Quantum Health — Website Data Export

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- Cochrane Breast Enlargement Evidence: Systematic Reviews, Implants & Protocols | Breast Enlargement Guide — /research/cochrane-breast-enlargement/
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About this website | Breast Enlargement Guide

Page: /about-us/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Independent education

About this website

Clear, balanced information about breast enlargement options — what works, what does not, and what to ask.

This website is an independent educational resource about breast enlargement. It aims to explain surgical and non-surgical options clearly, describe what the published evidence does — and does not — show, and set realistic expectations about benefits, limitations and risks.

There is no clinic, medical practice, doctor, team or physical address promoted here. Content is written to help readers prepare informed questions for a qualified, registered healthcare professional who can assess them in person.

Where possible, explanations are grounded in published research, peer-reviewed literature and guidance from recognised regulators and public-health bodies. Marketing claims are not accepted uncritically: terms like "regenerative", "natural" or "permanent" are explained by what evidence supports them.

This site does not sell treatments, products or procedures. Links to external sources are provided for verification. For decisions about your health, speak to an appropriately qualified clinician.

Learn and compare

Start with the evidence

Use the treatment guides, comparison page and research libraries to prepare questions before any consultation.

What to Know Before Choosing Breast Implants || Quantum Health

Page: /blogs/before-choosing-breast-implants/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Surgical education

What Should You Know Before Choosing Breast Implants?

Cover implant selection, informed consent, recovery, long-term monitoring and possible revision surgery.

Choosing breast implants is a medical decision as much as an aesthetic one. This guide outlines the areas worth understanding before you commit.

Implant selection

Implant selection depends on individual anatomy, product characteristics, surgical assessment and your preferences. There is no single “best” implant; what suits one person may not suit another.

Informed consent

Informed consent means understanding the procedure, its limitations, its risks and the alternatives — and having the opportunity to ask questions before deciding.

Recovery

Recovery varies according to the technique, individual healing and your surgeon’s advice. Your surgeon will provide a personalised return-to-activity plan.

Long-term monitoring

Breast implants are not lifetime devices. Additional operations may be needed, and complications can occur, so long-term monitoring is an important part of the decision.

Revision surgery

Some patients require revision surgery at some point. Understanding this possibility in advance is part of realistic planning.

Editorial disclaimer

This article is educational and does not provide a personal diagnosis or treatment recommendation. It is an editorial draft: an author and a suitably qualified clinical reviewer will be credited only once the article has actually been authored and reviewed.

Primary references consulted

- U.S. FDA — Breast implants (risks and long-term monitoring)
- American Society of Plastic Surgeons — Breast augmentation: implant types and safety (https://

www.plasticsurgery.org/cosmetic-procedures/breast-augmentation/implants)

Read next

Related articles

[Image: Abstract breast anatomy artwork for patient education]

Treatment guides

Surgical vs Non-Surgical Breast Enhancement: Understanding the Differences

Compare intended outcomes, limitations, recovery and risks — without assuming that all non-surgical treatments enlarge breast volume.

[Image: Breast-medical editorial illustration with gold and rose tissue contours]

Patient resources

Ten Questions to Ask at a Breast Enhancement Consultation

A practical checklist covering credentials, product information, suitability, risks, costs and aftercare.

Your journey starts with information

Keep exploring the evidence

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Breast Enlargement Without Surgery: What Works || Quantum Health

Page: /blogs/breast-enlargement-without-surgery-what-works/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Evidence and research

Breast Enlargement Without Surgery: An Evidence-Based Verdict

A method-by-method scorecard: what changes appearance, what adds real volume, and what to avoid.

More products are sold for breast enlargement without surgery than the evidence can support. A short scorecard — summarised from our full evidence guide — separates what works, what partly works, what is unproven, what does not work and what should be avoided outright.

The verdict pattern

Methods that work are either honest about being cosmetic — a well-fitted bra, padding, styling and posture change the look, not the breast, immediately and safely — or honest about being a procedure. Fat transfer moves real tissue through liposuction and is the best-supported route to lasting volume without an implant, but reviews average roughly 58% fat retention from about 300 mL injected: about one cup, with a complication rate that is not trivial.

Why most products fail

Breast tissue grows in response to internal hormonal signals, not to anything applied to the skin or worn over the chest. A cream or pill that truly delivered a growth signal would be an unregulated hormone treatment with effects throughout the body — so a harmless product is, almost by definition, one that does not enlarge the breast. Temporary fullness from suction cups, hormone-active ingredients or the menstrual cycle is swelling or cyclical change, not growth.

Add-ons that do not enlarge the breast

EMS devices contract the pectoral muscle beneath the breast; radiofrequency, ultrasound and laser warm skin for tightening; threads reposition tissue for a lift, not size. PRP and stem-cell enrichment sold as fat-graft boosters remain unproven — one review found enrichment could double the fat required while increasing retention only modestly.

What to avoid

The FDA recommends against using any injectable filler to increase breast size, and has warned that injectable silicone used for body contouring has caused disfigurement, blocked blood vessels and death, with problems appearing years later and no approved method of removal. Self-injection kits and unsupervised hormone products belong in the same category: decline them.

How to spot a false claim

“Clinically proven” often means an in-house survey. “Results in 30 days” usually means swelling or weight

change. “FDA registered” is facility registration, not product approval. Before spending money, search the product name with “warning” or “recall”, check your national regulator, and look for an independent published trial rather than a patent or testimonial.

A sensible first step

Get measured for a bra properly, work on posture, and decide what you actually want — size, shape, symmetry or confidence. If you want lasting change, take the written plan from a qualified, registered surgeon, compare more than one opinion, and decline anything that involves unapproved injections or unregulated products.

Editorial disclaimer

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Primary references consulted

- U.S. FDA — Dermal Fillers (Soft Tissue Fillers); 2017 safety communication on injectable silicone for body contouring; August 2025 advisory meeting executive summary
- Seth I, et al. — Autologous fat grafting in breast augmentation: a systematic review (35 studies, 3,757 women)
- Adipose-derived stem cell enrichment is counter-productive for primary aesthetic breast augmentation by autologous fat transfer: a systematic review. J Plast Reconstr Aesthet Surg, 2020
- “Bust enhancing” herbal products. Obstet Gynecol, 2003;101(6):1345–9
- American Society of Plastic Surgeons — 2025 plastic surgery statistics; breast augmentation patient information (https://www.plasticsurgery.org/news/plastic-surgery-statistics)

Read next

Related articles

[Image: Abstract breast anatomy artwork for patient education]

Treatment guides

Surgical vs Non-Surgical Breast Enhancement: Understanding the Differences

Compare intended outcomes, limitations, recovery and risks — without assuming that all non-surgical treatments enlarge breast volume.

[Image: Breast anatomy-inspired abstract contours with luminous rose highlights]

Evidence and research

PRP and Breast Enhancement: What Does the Evidence Say?

Explain the difference between researching a treatment and establishing its safety and effectiveness for a particular indication.

[Image: Abstract breast tissue illustration in burgundy, rose and champagne gold]

Safety and evidence

Breast Fillers, Sculptra and Biostimulators: Important Safety Questions

Explain product-specific indications and why facial aesthetic treatments cannot automatically be considered suitable for breast injections.

Your journey starts with information

Keep exploring the evidence

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Breast Fillers, Sculptra & Biostimulators: Safety || Quantum Health

Page: /blogs/breast-fillers-sculptra-and-biostimulators-safety-questions/

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Safety and evidence

Breast Fillers, Sculptra and Biostimulators: Important Safety Questions

Explain product-specific indications and why facial aesthetic treatments cannot automatically be considered suitable for breast injections.

Many injectable products are approved for specific facial indications. That approval does not extend to the breast.

Approval is specific

An approval for one indication does not imply suitability for another. Facial approval does not establish that a product is safe or appropriate for injection into the breast.

The breast is different

The breast is a different anatomical site with different considerations, and complications can be more difficult to manage. This is why regulators advise against using fillers to increase breast size.

Product-specific questions

Ask for the exact product, its approved indications, whether breast use is authorised in your jurisdiction, and how complications would be managed.

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- U.S. FDA — Dermal fillers and their approved uses
- U.S. FDA — Regenerative medicine and stem cell safety

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[Image: Abstract breast anatomy artwork for patient education]

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Breast Enlargement Education & Patient Resources | Quantum Health

Page: /blogs/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Knowledge for more confident decisions

Insights & Patient Resources

Clear, evidence-aware articles covering treatment options, safety, recovery and the questions worth asking before any decision.

Articles

Educational articles

Every article is an editorial draft pending clinical review; author and reviewer details are published only once verified.

[Image: Abstract breast anatomy artwork for patient education]

Treatment guides

Surgical vs Non-Surgical Breast Enhancement: Understanding the Differences

Compare intended outcomes, limitations, recovery and risks — without assuming that all non-surgical treatments enlarge breast volume.

[Image: Abstract breast-focused medical artwork in burgundy, rose and champagne gold]

Surgical education

What Should You Know Before Choosing Breast Implants?

Cover implant selection, informed consent, recovery, long-term monitoring and possible revision surgery.

[Image: Breast anatomy-inspired abstract contours with luminous rose highlights]

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[Image: Breast-medical editorial illustration with gold and rose tissue contours]

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A practical checklist covering credentials, product information, suitability, risks, costs and aftercare.

[Image: Bold abstract breast contour lines on a burgundy background]

Safety and evidence

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suitable for breast injections.

[Image: Breast anatomy-inspired abstract contours with luminous rose highlights]

Evidence and research

Breast Enlargement Without Surgery: An Evidence-Based Verdict

A method-by-method scorecard: what changes appearance, what adds real volume, and what to avoid.

Your journey starts with information

Prefer to see how the evidence fits together?

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

PRP and Breast Enhancement: Evidence Explained || Quantum Health

Page: /blogs/prp-and-breast-enhancement-what-the-evidence-says/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Evidence and research

PRP and Breast Enhancement: What Does the Evidence Say?

Explain the difference between researching a treatment and establishing its safety and effectiveness for a particular indication.

PRP is prepared from a patient’s own blood to concentrate platelets. The key question is not what PRP is, but what the evidence supports for the specific use being advertised.

How PRP is prepared

PRP is prepared by processing a sample of the patient’s own blood to concentrate platelets. A preparation intended for one indication is not automatically appropriate for another.

Research vs. established use

There is an important difference between studying a treatment and establishing that it is safe and effective for a particular purpose. Claims of breast enhancement should be assessed against appropriate clinical evidence for the precise procedure offered.

What to ask

Ask what evidence supports the specific use, what the known risks are, and what regulatory requirements apply to that intended use in your jurisdiction.

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Primary references consulted

- U.S. FDA — Dermal fillers and their approved uses
- U.S. FDA — Regenerative medicine and stem cell safety

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Related articles

[Image: Abstract breast anatomy artwork for patient education]

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[Image: Abstract breast tissue illustration in burgundy, rose and champagne gold]

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Surgical vs Non-Surgical Breast Enhancement || Quantum Health

Page: /blogs/surgical-vs-non-surgical-breast-enhancement/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Treatment guides

Surgical vs Non-Surgical Breast Enhancement: Understanding the Differences

Compare intended outcomes, limitations, recovery and risks — without assuming that all non-surgical treatments enlarge breast volume.

The phrase “breast enhancement” covers very different procedures with very different goals. Understanding those differences is the first step toward an informed decision.

Why the distinction matters

Not every treatment marketed as “non-surgical breast enlargement” has demonstrated a reliable increase in breast volume. Surgical and non-surgical approaches differ in their intended effect, their evidence base, their risks and their recovery requirements. Treating them as interchangeable can lead to unrealistic expectations.

Intended outcome

Surgical augmentation is designed to increase breast volume or change breast shape. Some non-surgical treatments target skin quality or provide a different aesthetic effect rather than a meaningful increase in volume. Others are marketed for volume but lack robust supporting evidence for that specific use.

Evidence and regulatory status

The evidence base differs sharply. Dermal fillers are not approved for breast enlargement, and regulators advise against using them for this purpose. Regenerative and biostimulatory products require careful scrutiny of their evidence and regulatory status.

Recovery and risk

“Non-surgical” does not mean risk-free. Any procedure carries some risk, and the consequences of complications can differ substantially between approaches. Surgery carries surgical and implant-related risks and may involve future procedures.

How to decide

The appropriate option depends on your health, anatomy, expectations and risk tolerance, and on the evidence for the intended treatment. That assessment should be made by an appropriately qualified clinician, not from advertising alone.

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actually been authored and reviewed.

Primary references consulted

- U.S. FDA — Dermal Fillers (approved uses and safety communications)
- U.S. FDA — Regenerative medicine and stem cell safety
- American Society of Plastic Surgeons — Breast augmentation and fat transfer patient information (https://www.plasticsurgery.org/cosmetic-procedures/breast-augmentation)

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Related articles

[Image: Breast anatomy-inspired abstract contours with luminous rose highlights]

Evidence and research

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Explain the difference between researching a treatment and establishing its safety and effectiveness for a particular indication.

[Image: Breast-medical editorial illustration with gold and rose tissue contours]

Patient resources

Ten Questions to Ask at a Breast Enhancement Consultation

A practical checklist covering credentials, product information, suitability, risks, costs and aftercare.

Your journey starts with information

Keep exploring the evidence

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Ten Questions to Ask at a Consultation || Quantum Health

Page: /blogs/ten-questions-to-ask-at-a-breast-enhancement-consultation/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

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Ten Questions to Ask at a Breast Enhancement Consultation

A practical checklist covering credentials, product information, suitability, risks, costs and aftercare.

A good consultation is a two-way conversation. These questions can help you get the information you need to decide.

1–3: Credentials and experience

What are your qualifications and registration? Which specific breast procedures do you perform, and how often? Who will carry out my procedure and follow-up?

4–6: The proposed treatment

What exactly are you recommending, and why? What are the approved indications for that product or procedure? What can it realistically achieve — and what can it not?

7–8: Risks and alternatives

What are the risks and possible complications, and how would they be managed? What alternatives exist, including doing nothing?

9–10: Cost and aftercare

What is the full cost, including any revision or follow-up? What aftercare and follow-up will I receive, and when should I seek medical attention?

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Primary references consulted

- U.S. FDA — Dermal fillers and their approved uses
- U.S. FDA — Breast implants
- American Society of Plastic Surgeons — Questions to ask your plastic surgeon (https://www.plasticsurgery.org/cosmetic-procedures/breast-augmentation/questions)

Read next

Related articles

[Image: Abstract breast anatomy artwork for patient education]

Treatment guides

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Compare intended outcomes, limitations, recovery and risks — without assuming that all non-surgical treatments enlarge breast volume.

[Image: Stylised breast-medical editorial illustration with luminous tissue layers]

Surgical education

What Should You Know Before Choosing Breast Implants?

Cover implant selection, informed consent, recovery, long-term monitoring and possible revision surgery.

Your journey starts with information

Keep exploring the evidence

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Breast Anatomy Explained: Tissue, Ligaments and Blood Supply

Page: /breast-anatomy/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Foundations

Breast Anatomy: The Structures That Matter

Understanding the structures of the breast explains why some treatments can add volume, why sagging happens, and why injecting or operating carries the risks it does.

The breast is not a simple sac of tissue. It is a layered structure whose behaviour — how it changes with weight and hormones, how it sags, and how it responds to surgery or injection — follows from what it is made of and how it is supplied with blood and nerve.

Structures

The layers and what each one does

Structure What it is Why it matters Skin & subcutaneous fat The outer envelope and the fat immediately beneath it Determines the covering of the breast and where surgery marks fall Glandular tissue Lobules and ducts that produce and carry milk The functional tissue; its proportion to fat varies greatly between people Fatty tissue Adipose tissue distributed around the glandular tissue A large part of breast volume; follows overall body fat and can be moved by fat transfer Cooper’s ligaments Fibrous bands anchoring the breast to the chest wall and skin Support the breast; stretching with age, weight change and pregnancy contributes to sagging Pectoral muscles Chest muscles lying under the breast There is no muscle in the breast itself — so training builds the wall, not the breast Blood supply Arteries feeding the breast from several directions Critical to fat-graft survival and to safe surgery; a blocked vessel by injected material can be catastrophic Nerves Sensory and autonomic nerves to breast and nipple Their position explains why surgery and procedures can change nipple and skin sensation Lymph drainage Vessels draining toward the axilla (armpit) and internal mammary chain Relevant to cancer detection and to interpreting injected material on imaging

Why anatomy shapes every treatment decision

Existing tissue. A breast with little natural tissue cannot hide a large implant or supply much fat for reabsorption; a breast with a lot of tissue behaves differently again. This is why implant selection and fat-transfer volumes are individual decisions, not catalogue choices.

Skin quality and laxity. Skin and the suspensory ligaments determine how well a breast holds its shape. Stretched skin after pregnancy or weight change is one reason a lift may be discussed alongside augmentation.

Blood supply. Fat survives only where the tissue around it can supply blood. Poor blood supply is a central reason a proportion of transferred fat does not survive, and why large-volume grafting is unreliable. It is also why any injected material that obstructs a vessel is dangerous.

Sensation and breastfeeding. Nerve pathways to the nipple pass through the breast. Depending on the technique, surgery and some procedures can affect nipple sensation or future breastfeeding, which is why these are discussed before consent.

Anatomy is why “non-surgical” is not automatically “safe”

The breast is a blood-rich, nerve-dense area that drains to the lymph nodes. Material injected into it can migrate, obstruct vessels, provoke inflammation and complicate cancer screening. That is the anatomical reason

regulators advise against injecting fillers or silicone to enlarge the breasts — not merely a labelling matter.

Putting it in context

This page gives a general anatomical overview. Individual anatomy varies widely, and how any structure affects your own options can only be assessed in person by a qualified clinician. For the evidence context, see the evidence guide and the research hub .

Your journey starts with information

See how anatomy shapes the treatment options

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Breast Enhancement Options Compared | Breast Enlargement Guide

Page: /breast-enhancement-comparison/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Decision guide

Breast Enhancement Comparison: Intended Effect vs. Claims

This comparison helps distinguish a procedure’s intended effect from claims made in advertising. It does not suggest that a non-surgical procedure is automatically safer than surgery.

At a glance

Intended effect, primary consideration and key limitation

Option Primary consideration Important limitation Dermal fillers Product-specific approved indications Not FDA-approved for breast enlargement PRP Evidence for the particular intended use Should not be promoted as proven breast enlargement Threads & mono threads Product-specific aesthetic indications Not established substitutes for breast augmentation Sculptra / biostimulators Product-specific authorised uses Do not assume suitability for breast-volume enhancement Stem-cell approaches Evidence and regulatory status of the exact product Many marketed regenerative uses remain unapproved or investigational Silicone breast implants Surgical increase in breast volume Surgery, implant-related risks and potential future procedures Fat transfer Uses the patient’s own fat for augmentation Variable fat retention and a limited, individualised result
Choosing between options is not a matter of finding the “safest” label. It requires understanding your goals, your anatomy and health, the evidence for each intended effect, and the risks you are prepared to accept. A consultation with an appropriately qualified clinician is the appropriate way to reach an individualised view.

Your journey starts with information

Not sure which option to consider?

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Breast Enlargement Research & PubMed Evidence | Breast Enlargement Guide

Page: /breast-enlargement-research/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Research library · PubMed

Breast Enlargement Research: What the Published Evidence Shows

Breast enlargement studies indexed in PubMed — surgical and non-surgical — summarised by topic, with a link to every original record.

This page collects peer-reviewed research on breast enlargement — surgical and non-surgical — indexed in PubMed, the US National Library of Medicine's database of biomedical literature. In October 2026 we searched PubMed using title and abstract terms covering breast augmentation, breast enlargement, fat transfer and autologous fat grafting, non-surgical enlargement, breast implants and breast fillers. Of 77 unique records retrieved, the most relevant are summarised here, grouped by topic, with a link to each original PubMed record.

What the literature shows in brief: breast augmentation with implants remains one of the most frequently performed aesthetic operations worldwide, with predictable volume but real complication and revision rates. Autologous fat grafting is the best-supported route to lasting volume without an implant, but the gain is modest and complications occur in roughly a quarter of cases in the largest systematic review. External vacuum expansion has produced true tissue growth in small studies, but demands weeks of daily device wear. Injectable fillers are not approved for breast enlargement, and published case series document serious complications. Creams, pills and most devices have no credible supporting trials.

How this page was compiled

Searches were run on PubMed (National Library of Medicine, USA) in October 2026 using title/abstract terms: "breast augmentation", "breast enlargement", ("fat transfer" OR "autologous fat grafting" OR "fat grafting") AND breast augmentation, ("non-surgical" OR filler OR PRP OR "platelet-rich plasma") AND breast enlargement, and breast implants AND (augmentation OR enlargement). 77 unique records were retrieved; the studies most relevant to breast enlargement are summarised below. Summaries use abstract-level data — consult the full article and current guidelines before drawing clinical conclusions. Inclusion here is not an endorsement of any study or treatment.

Evidence at a glance

The headline findings, by question

Each row links to the PubMed record behind the finding.

Question What the published evidence shows Key studies Does fat transfer enlarge breasts? Yes — modestly. The largest systematic review (35 studies, 3,757 women) reported an average 58% fat retention from about 300 mL injected per breast — roughly one cup — with a 27.8% overall complication rate and 92% average patient satisfaction. Seth 2024 (PMID 37166041) Is fat transfer better than implants? The only randomised trial identified found implants gave more evident and durable augmentation (87.5% vs 70% "excellent" aesthetic outcomes), while fat grafting produced more natural-looking results and better correction of chest-wall deformity. Gentile 2026 (PMID 40908320) Can a vacuum device enlarge breasts without surgery? A 2000 study reported a stable 55% average size increase after 10 weeks of daily external expansion (10–12 h/day); MRI confirmed growth of fat and fibroglandular tissue. Evidence remains limited and largely developer-led. Khouri 2000 (PMID

10845308); Kardorff 2004 (PMID 16285326) Do breast fillers work — and are they safe? Hyaluronic acid gel was briefly authorised for breast use in Europe (2008), then withdrawn worldwide after side effects. A 31-patient complication series found 88.9% of conservatively treated aquafilling cases needed reoperation. Regulators advise against fillers for breast enlargement. Siebert 2014 (PMID 24507861); Gierej 2023 (PMID 37721627); Jeyaratnam 2021 (PMID 34849315) How often do implants need revision? Reported revision rates after primary augmentation are as high as 36%, most commonly for capsular contracture, implant malposition and ptosis of the ageing breast. Denney 2021 (PMID 34121945) Is “breast implant illness” established? Systemic symptoms attributed to implants are widely reported, but a 2024 Danish review notes the correlation with implants is not established, evidence is limited, and it is not a recognised diagnosis. Borhani-Khomani 2024 (PMID 38808767) How common is augmentation worldwide? ISAPS 14-year global data show breast augmentation consistently among the top five surgical aesthetic procedures. Implant removal has been the fastest-growing procedure since 2015 (+46.3% over five years). Triana 2024 (PMID 39103642)
Study figures describe the groups studied and may not apply to any individual. Evidence and regulatory positions change — check the linked records and current regulator guidance before acting.

Fat transfer (autologous fat grafting)

Fat transfer removes fat by liposuction from another body area and injects it into the breast. It is the best-documented route to real, lasting volume without an implant — but the gain is modest, and the evidence base, while growing, is still dominated by retrospective series rather than controlled trials.

Fat transfer studies

What fat-grafting research reports

Study Design What it found Seth I, et al. 2024 — Plast Reconstr Surg (PMID 37166041) Systematic review: 35 studies, 3,757 women Average 300 mL injected per breast; 58% volume retention (range 44–83%); 27.8% overall complication rate, fat necrosis the most common (43.7% of complications); radiologic fat necrosis 9.4%, calcification 1.2%; 92% average satisfaction at one year. Gentile P. 2026 — Aesthetic Plast Surg (PMID 40908320) Randomised open-label trial: 95 implants vs 90 lipofilling 87.5% of implant patients vs 70% of lipofilling patients had excellent aesthetic outcomes at 12 months. Implant results were significantly more durable; lipofilling results were judged more natural and improved pectus excavatum. Both were safe and effective. Voglimacci M, et al. 2015 — Aesthetic Surg J (PMID 25908697) Systematic review: 42 articles Evidence mostly low level (one level-2 study). Two cancers among 2,023 patients (0.09%) with insufficient follow-up for long-term conclusions. Concludes registries with proper follow-up are needed. Leopardi D, et al. 2014 — ANZ J Surg (PMID 23656507) Systematic review vs implant comparators No comparative studies were found. Fat necrosis, cysts and lumps were the most common complications; reabsorption occurs mainly in the first 12 months; efficacy could not be determined from the available evidence. Hivernaud V, et al. 2015 — Aesthetic Plast Surg (PMID 26085223) Technique review High resorption remains the central problem of large-volume grafting; refinements in harvesting and graft preparation aim to improve fat viability and long-term success. Coleman SR, Saboeiro AP. 2015 — Clin Plast Surg (PMID 26116935) Technique review The 2009 ASPS Fat Graft Task Force concluded fat grafting may be considered for breast augmentation, with results dependent on technique and surgeon expertise. Finzi E. 2003 — Dermatol Surg (PMID 12930335) Retrospective record review Breast enlargement occurred in 43% of patients after tumescent liposuction of other body areas; 30% gained one cup size or more — an incidental, redistribution-based effect. Seth I, et al. 2023 — Gland Surg (PMID 36761486) Method paper: hybrid augmentation Proposes a preoperative formula to estimate fat-graft volume when combining implants with lipofilling (hybrid breast augmentation).

Non-surgical enlargement: external expansion

External vacuum expansion devices (bra-like systems worn for hours a day) aim to grow tissue through controlled mechanical stretch. Small studies report measurable growth; the regimens are demanding, and critics have questioned measurement consistency.

Expansion studies

External expansion: the published record

Study Design What it found Khouri RK, et al. 2000 — Plast Reconstr Surg (PMID 10845308) Single-group study: 17 women (12 completed) A brassiere-like system applying 20 mmHg per breast, 10–12 h/day for 10 weeks: peak increase 98%; after recoil, a stable long-term increase of 55% (range 15–115%). MRI confirmed proportionate growth of adipose and fibroglandular tissue. No adverse events recorded. Kardorff B, et al. 2004 — JDDG (PMID 16285326) German case report using the same protocol Chest circumference at the nipple level remained 4 cm above baseline at three months after oedema resolved. Side effects were limited to transient irritant contact dermatitis and folliculitis. He Y, et al. 2017 — Plast Reconstr Surg (PMID 28350663) Preclinical/technique study Combining minimally invasive tissue dissection with external volume expansion generated far more adipose tissue than expansion alone, mediated by release of cell-contact inhibition. Tebbetts JB. 2001 — Plast Reconstr Surg (PMID 11379562) Critical review Argued that claimed non-surgical enlargement largely reflects measurement variability and soft-tissue change rather than reliable true tissue growth — a caution against overstating the early expansion data. Uyulmaz S, et al. 2020 — Praxis (PMID 33109001) Clinical review Reviews implant-free cosmetic augmentation: mastopexy for optical lift and lipofilling for volume; large-volume or high-goal augmentation with fat alone remains limited.

Fillers and injectable products

No injectable filler is approved for breast enlargement by major regulators. The published literature documents what happens when fillers are used in the breast: product withdrawal, complication series and reoperation.

Filler safety evidence

What the filler studies report

Study Design What it found Siebert T, et al. 2014 — Ann Chir Plast Esthet (PMID 24507861) Literature review (Macrolane) Hyaluronic acid gel (Macrolane) received European authorisation for breast augmentation in 2008, then was withdrawn from the worldwide breast-augmentation market after numerous side-effect controversies. Gierej P, et al. 2023 — Aesthetic Plast Surg (PMID 37721627) Observational cohort: 31 women with aquafilling-gel complications 88.9% of patients treated conservatively required reoperation, vs 22.7% after radical surgery. Needle aspiration was ineffective and associated with infection (75% positive cultures). More filler injected correlated with more symptoms. Jeyaratnam S, Joethy J. 2021 — PRS Global Open (PMID 34849315) Case-based review Documents the dangers of filler breast augmentation, consistent with regulator advice against the practice.

Implant augmentation: outcomes, complications and revision

Implant surgery has the largest evidence base of any augmentation method. The studies below cover complication prevention, revision burden, device history, imaging and patient-reported systemic symptoms.

Implant studies

Implant augmentation: the study data

Study Design What it found Denney BD, et al. 2021 — Semin Plast Surg (PMID 34121945) Review Revision rates after primary augmentation remain as high as 36%; the commonest causes are capsular contracture, implant malposition and ptosis of the ageing breast. Nava MB, et al. 2017 — Gland Surg (PMID 28497025) Evidence review No high-level evidence identifies a single best technique or implant; complication reduction follows established surgical principles rather than device fashion. Nahabedian MY. 2019 — Plast Reconstr Surg (PMID 31246764) Review of form-stable implants Reviews diagnosis and management of complications with modern cohesive-gel implants; no implant lasts forever, though devices remain safe and effective per clinical and epidemiologic studies. Kaoutzanis C, et al. 2019 — Semin Plast Surg (PMID 31632203) Historical review Traces implant evolution since 1962, including the silicone-gel controversies, and how successive generations improved safety and aesthetics. Correia-Sá I, et al. 2013 — Aesthetic Plast Surg (PMID 23943050) Case report with histology PIP implants showed increased rupture and leakage risk; documents clinical, histologic and

microbiologic findings after rupture, including siliconoma and lymph-node involvement. Monstrey S, et al. 2004 — Plast Reconstr Surg (PMID 15108875) Case series: 13 patients Trilucent (soybean-oil-filled) implants failed because of lipid-oxidation metabolites; UK and Belgian authorities advised removal — a documented device-withdrawal precedent. Simádo TS. 2021 — Plast Reconstr Surg (PMID 33885442) Clinical algorithm Galactorrhea and galactocele after breast augmentation: a diagnosis and treatment algorithm. Green LA, et al. 2018 — Clin Imaging (PMID 28918365) Imaging review Breast-cancer screening must continue after augmentation; radiologists need familiarity with surgical techniques, implant appearances and MRI findings. Faguy K. 2018 — Radiol Technol (PMID 29793920) Imaging review Catalogues adverse effects and imaging appearances of breast implants for technologists and radiologists. Borhani-Khomani K, et al. 2024 — Ugeskr Læger (PMID 38808767) Danish review Self-reported systemic symptoms attributed to implants (“breast implant illness”): the correlation is not established, evidence is limited, and the condition is not currently a recognised diagnosis. Luan J. 2023 — Clin Plast Surg (PMID 36396254) Technique review Endoscopic-assisted transaxillary augmentation: the axillary approach remains preferred by many patients in Asia; endoscopy improves control and stability of the result. Bekeny JC, et al. 2020 — Gland Surg (PMID 32775269) Review (transfeminine augmentation) Augmentation for transfeminine patients is technically similar to that for cisgender peers, with equivalent complication rates and documented quality-of-life benefits. Holst-Albrechtsen S, et al. 2018 — Ugeskr Læger (PMID 29886889) Clinical study Post-operative infections after cosmetic surgery abroad (“cosmetic tourism”) — treatment abroad carries identifiable infection risk.

Global trends and contemporary benchmarks

Large registry analyses show how often augmentation is performed, how preferences are shifting, and where the field is heading.

Trends

What the registry and benchmark studies report

Study Design What it found Triana L, et al. 2024 — Aesthetic Plast Surg (PMID 39103642) ISAPS global statistics, 2010–2023 34.9 million aesthetic procedures worldwide in 2023. Breast augmentation has remained consistently in the top five surgical procedures for 14 years, with a first decline in the most recent year; implant removal is the fastest-growing procedure since 2015 (+46.3% over five years). Glicksman CA. 2026 — Clin Plast Surg (PMID 41826039) Benchmarks review Contemporary benchmarks in breast augmentation surgery — current reference points for planning and outcomes. Lhuaire M, et al. 2019 — Plast Reconstr Surg (PMID 31764702) Technique paper Minimally invasive inframammary breast augmentation — approaches aimed at smaller, hidden incisions. Lin Y, et al. 2022 — Aesthetic Plast Surg (PMID 33403418) Anthropometric study Body and breast measurements in augmentation planning — objective sizing data supporting implant selection. Questions

About this research page

What is PubMed, and why use it?

PubMed is the US National Library of Medicine’s free index of biomedical literature, covering thousands of peer-reviewed journals. Searching it gives a structured, reproducible view of what has actually been studied — rather than what is advertised.

Does listing a study mean we recommend its findings?

No. Listing means the study is part of the published evidence base. Individual studies vary in quality — a small case report carries far less weight than a systematic review or randomised trial. Nothing on this page is a recommendation for any individual.

Why do study findings sometimes disagree?

Different designs, patient groups, surgical techniques, measurement methods and eras produce different results. Systematic reviews that pool many studies sit at the top of the evidence ladder; single-centre case series sit lower.

How current is this page?

The PubMed search was run in October 2026. New studies are indexed continuously — check PubMed for the latest work, and look for updated systematic reviews and regulator statements.

Where can I read the full articles?

Each link opens the study's PubMed record with its abstract. Many full texts sit behind journal subscriptions; ask a librarian, university library or your clinician for access, and treat any claim you cannot verify with caution.

Educational information only — not medical advice

This page summarises published research for general education. It cannot account for your health, anatomy or circumstances, and it does not recommend any treatment. Study figures may not transfer to individuals. Decisions about breast enlargement should be made with a qualified, registered clinician after proper assessment. If you notice a new breast lump or any urgent symptom, seek medical care promptly.

Your journey starts with information

Want to discuss what the evidence means for you?

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Breast Enlargement Safety: Candidacy, Risks and Warning Signs

Page: /breast-enlargement-safety/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Safety

Breast Enlargement Safety

Safety is not a property of a treatment label; it is the balance of benefit, risk and suitability for one person. This page brings the safety points from every option into one place.

“Non-surgical” describes a method, not a risk level. A cream carries almost no medical risk but also does nothing; an injectable can be marketed as non-surgical and still cause serious harm; surgery carries the risks of any operation. Safety means matching the smallest effective intervention to your actual goal, after honest assessment.

Candidacy

Factors that shape whether a procedure is appropriate

General health

Medical conditions, medications and allergies can affect whether a procedure is suitable and how healing proceeds.

Timing

Pregnancy, breastfeeding and significant planned weight change all argue for waiting.

Anatomy & tissue

Existing tissue, skin quality and chest-wall proportions determine what is realistically achievable.

Age

Breasts may keep developing into the late teens and early twenties; most professional guidance advises against enlargement procedures for minors.

Expectations

Realistic expectations — and a willingness to accept that revision may be needed — are part of candidacy.

Contraindications

Active infection, certain uncontrolled conditions and pregnancy are typical reasons to postpone or decline a procedure.

Risk by route

What can go wrong, by option

Route Main risks Severity Bras, padding, styling Skin irritation from adhesive products Very low Creams and pills Allergic reactions; undeclared or hormone-active ingredients Low, but no benefit Suction / vacuum devices Bruising, broken vessels, skin damage Low to moderate Fat transfer Fat necrosis, oil cysts, calcification, infection, asymmetry; complications reported in about a quarter of cases in the largest review Moderate External

expansion + fat transfer Everything above plus skin problems and rare serious events from prolonged device wear Moderate Implants (surgery) Capsular contracture, rupture, ALCL, infection, sensation change, revision surgery Moderate to serious Injectable fillers in the breast Nodules, migration, infection, imaging interference, difficulty of removal; not approved for this use High Liquid silicone / unknown injectables Disfigurement, embolism, stroke or death; problems may appear years later Very high
Figures describe the groups studied and may not apply to any individual. See the evidence guide for the data behind the non-surgical rows.

Warning signs — seek help promptly

- Fever, spreading redness, pus or worsening pain after any procedure — possible infection; contact your clinician the same day.
- Pale, bluish, dusky or dark skin, or blisters — may mean reduced blood supply; seek urgent care.
- Chest pain, breathlessness or collapse — possible blood clot; call emergency services.
- Sudden vision change, severe headache or weakness after an injection — possible blocked vessel; call emergency services.
- A new lump, hardness or skin change — have it examined and imaged.

Questions

Safety FAQs

Is non-surgical always safer than surgery?

No. A cream is low-risk but ineffective; unapproved breast filler is “non-surgical” yet linked to serious harm; surgery carries real but better-understood risks. Safety depends on the specific product, the indication and who performs it.

How do I know if a clinic is safe?

Check registration and credentials, insist on a written plan and revision policy, ask who performs the procedure and where, and treat pressure or guarantees as red flags. See choosing a surgeon .

Are there signs I should not have a procedure at all?

Yes. Active infection, pregnancy or breastfeeding, some uncontrolled medical conditions, and unrealistic expectations are typical reasons to postpone or decline. A good clinician will say so.

Regulator sources

U.S. Food and Drug Administration (FDA) — www.fda.gov/

- Breast Implants (Regulatory hub)
- Types of Breast Implants (Regulatory page)
- Risks and Complications of Breast Implants (Regulatory page)
- Things to Consider Before Getting Breast Implants (Patient decision guide)
- Breast Implant Surgery (Regulatory page)
- Breast Implants — Certain Labeling Recommendations to Improve Patient Communication (FDA guidance (2020))
- …and 10 more in the full library.

National Health Service (NHS, UK) — www.nhs.uk/

- Breast enlargement (implants) (Patient information page)
- Is a cosmetic procedure right for me? (Advice page)
- Before you have a cosmetic procedure (Advice page)
- Choosing who will do your cosmetic procedure (Advice page)

- Is cosmetic surgery available on the NHS? (Advice page)
- Cosmetic surgery abroad (Advice page)

Educational information only

This page is general education, not a personal assessment. Suitability, risks and outcomes vary and require evaluation by an appropriately qualified clinician.

Your journey starts with information

Take your safety questions to a qualified clinician

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Breast Enlargement Without Surgery: What Works | Evidence Guide

Page: /breast-enlargement-without-surgery/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Evidence guide · Scorecard

Breast Enlargement Without Surgery: What Works and What Doesn’t

What works, what doesn’t — a verdict for every popular method, with the evidence, the risks and the red flags behind it.

This guide answers one question: which non-surgical ways of enlarging the breasts actually work, and which do not? Each method gets a verdict, the evidence behind it and the risks. It draws on regulator statements and published medical reviews, and it is general education — not a recommendation for any individual.

The short version: methods that change how the breasts look — a well-fitted bra, padding and styling — work immediately and safely. The only approaches shown to add real, lasting volume without an implant are fat-based: fat transfer, with or without an external device worn beforehand. Those give a modest gain and still involve a procedure. Nearly everything else sold to enlarge breasts, including creams, pills, massage and suction cups, has no reliable evidence behind it, and injectable fillers and silicone are actively dangerous.

How to read the verdicts

What the five verdicts mean

Verdict Meaning WORKS Delivers what it claims — either a visual change (bras, styling) or a measurable, lasting volume gain (fat transfer) — with risks that are understood. PARTLY Delivers part of the promise, for example a firmer-looking chest or promising but demanding results, but not a simple route to a bigger breast. UNPROVEN Plausible or widely used, but good evidence for breast enlargement is missing or mixed. DOESN’T WORK No reliable evidence of lasting enlargement, or the effect is only temporary swelling. AVOID Not approved for this use and linked to serious harm. The risks outweigh any possible benefit.
The scorecard

More than 20 popular methods, judged

Each row gives a verdict, what the method actually does, and the bottom line.

Method Verdict What it actually does Bottom line Correctly fitted bra WORKS (look) Changes shape and apparent size immediately Best first step. No medical risk. Padded, push-up and adhesive bras; inserts WORKS (look) Adds roughly a cup visually while worn Reversible and inexpensive. Clothing, styling and make-up contouring WORKS (look) Creates fullness through necklines, layers and highlight Free to try. Posture training WORKS (look) Opens the chest and removes a stooped look Visible difference, no cost. Chest strength training PARTLY Builds the muscle under the breast; firmer chest wall Does not enlarge breast tissue. Weight gain PARTLY Raises breast fat but cannot be targeted Unpredictable; not advised for this goal. Fat transfer WORKS (modest) Adds your own fat; a review averaged about 58% retained Roughly one cup; it is a surgical procedure. External expansion plus fat transfer PARTLY Stretches tissue first so more fat survives Promising, but demanding and mostly developer-led data. PRP or stem-cell (SVF) add-ons UNPROVEN Claimed to help fat survive Debated; may cut usable volume. Muscle stimulators (EMS) DOESN’T WORK Contracts the chest muscle beneath Cannot enlarge breast tissue. Radiofrequency, ultrasound, laser DOESN’T WORK Warms skin

for tightening Tightening, not volume. Thread lifts DOESN'T WORK Repositions tissue Lift, not size. Creams and lotions DOESN'T WORK Moisturise the skin surface Cannot grow glandular or fatty tissue. Herbal pills and teas DOESN'T WORK Contain plant compounds with hormonal activity No clinical trials; safety concerns. Foods and diets (soy, flax, milk, papaya) DOESN'T WORK Nutrition only No effect on breast volume. Massage DOESN'T WORK Relaxes and warms the skin No growth. Suction and vacuum cups DOESN'T WORK Draws fluid into the tissue Temporary swelling; bruising risk. Hypnosis, subliminal audio, magnets, patches, acupuncture DOESN'T WORK No credible mechanism No credible evidence. Hyaluronic acid, CaHA or PLLA fillers in the breast AVOID Inject gel into the breast area FDA recommends against; imaging problems. Liquid silicone, oils and unknown materials AVOID Inject permanent or unknown material Linked to disfigurement and death. Self-injection and needle-free filler devices AVOID Unregulated delivery Not approved; serious harm reported. Hormone products without supervision AVOID Alter hormone levels Not a cosmetic route; real health risks. Implants (surgical, for comparison) WORKS (surgical) Place a prosthesis through an incision Reliable size, but outside the scope of this guide.
The pattern to remember: methods that work are either honest about being cosmetic — they change the look, not the breast — or honest about being a procedure — they move real tissue, with real risks. Methods that promise growth without either of these are the ones that fail.

How we judged whether a method works

Four questions were applied to every method: Does it add measurable volume? — not a feeling of fullness or a better-fitting bra, but more breast. Does it last? — weeks of swelling is not enlargement. Is it safe enough for the benefit? — a small gain is not worth a serious risk. Is the evidence independent and well designed? — who ran the study, who paid for it, and were the results compared with a fair control? Systematic reviews sit at the top of the evidence ladder; manufacturer surveys, patents and testimonials sit at the bottom.

Why most methods fail: how breast tissue actually grows

The breast's duct system and the fat around it develop mainly in response to hormones, above all oestrogen, during puberty, with further changes in pregnancy and breastfeeding. In adult life, breast volume mostly follows body fat, the hormonal cycle, pregnancy and age. The signal that builds breast tissue is internal and hormonal, and nothing applied to the skin or worn over the chest sends that signal.

Only three routes are known to add tissue: hormonal stimulation (which belongs under medical supervision for medical reasons only, not cosmetics), fat transfer (your own fat moved by a qualified surgeon — modest and surgical), and implants (surgery, outside this guide). Creams, pills, massage, cups and devices have no established mechanism for adding tissue at all.

Why creams and pills cannot do it

To grow breast tissue, a cream or pill would have to deliver a growth signal to deep tissue. The only substances known to do that are hormones, which are drugs with effects throughout the body. A product that truly worked in that way would not be a harmless cosmetic — it would be an unregulated hormone treatment. A product that is harmless is, almost by definition, one that does not do the job.

A quick test: swelling or growth?

- If fullness fades within hours or days, it was fluid, swelling or a hormonal cycle effect, not growth.
- If it appears and disappears with your cycle, it is a normal cyclical change.
- If it follows weight gain, the fat changed — and that would happen without the product.
- Real, lasting added volume appears only after procedures that place tissue or material in the breast, or after hormonal development.

Works for appearance: bras, styling and posture

These options do not change the breast itself. They change how it looks and sits, and they do it immediately, cheaply and without medical risk. For many people who ask about enlargement, they are the most effective and least regrettable place to start. Fitting professionals often report that many people wear a band that is too loose and a cup that is too small — a bra that fits properly lifts and supports the breast, which projects the chest

forward and changes the shape and apparent size more than any pill or cream.

Bra styles

Styles and what they do

Style Effect Best for Padded T-shirt bra Smooth shape and modest added fullness Everyday wear under fitted tops Push-up bra Lifts and brings the breasts together; often reads as about a cup larger Cleavage, low necklines Balconette Lifts the upper curve into a rounded shape Square and wide necklines Plunge Brings the breasts together at the centre Deep necklines Adhesive or stick-on Lifts and shapes without straps or a band Backless and strapless outfits; may irritate sensitive skin Silicone or foam inserts Adds volume inside a bra or swimwear Asymmetry or extra fullness; keep clean and dry
Necklines such as boat, sweetheart and wrap styles draw the eye across the chest. Ruching, gathers, pleats, prints and layering add visual volume at the bust line. Rounded shoulders and a stooped upper back hide the chest — drawing the shoulders gently back and down gives an immediate difference in how the chest looks.

Partly works: strength training and weight

The breast is made of glandular tissue, fat and connective tissue, with skin around it. There is no muscle inside the breast. The pectoral muscles lie underneath, so chest exercises build the platform the breast sits on, not the breast. Training can firm and broaden the chest wall, improve posture and shoulder support, and make the breast sit slightly higher and fuller in clothes. It cannot enlarge glandular or fatty breast tissue, reverse sagging caused by stretched skin and ligaments, or target fat gain or loss in the breasts.

Because breasts contain a lot of fat, gaining weight often increases size — but you cannot choose where weight is gained, and gaining weight purely for this goal carries its own health trade-offs, so it is not a sensible strategy. The reverse also holds: fat loss from heavy training or dieting can reduce breast size, and rapid weight change can leave skin looser.

Chest routine

Suggested exercises if you train

Exercise Suggested volume Notes Incline push-ups (hands on a bench or table) 3 sets of 8–12 Lower the incline as you get stronger. Dumbbell chest press 3 sets of 8–12 Choose a weight that makes the last two reps challenging. Dumbbell chest fly 2 sets of 10–12 Light weights, elbows slightly bent. Seated or bent-over row 3 sets of 10–12 Balances the chest work and supports posture.
Train two to three non-consecutive days a week and warm up first. If you have a shoulder, wrist or heart condition, or are new to exercise, ask a doctor or qualified trainer first.

Works modestly: fat transfer

Fat transfer (also called lipofilling or autologous fat grafting) removes fat from another body area by liposuction, processes it, and injects it into the breast in small amounts. It leaves no implant. It is often marketed as non-surgical, but because it involves liposuction and, usually, sedation or anaesthesia, it is better described as minimally invasive surgery. It is the best-supported route to real, lasting volume without an implant — but the gain is modest and unpredictable, complications are not rare, and a surgeon who quotes a range rather than a guarantee is giving you honest information.

What the evidence shows

Fat transfer: the published figures

Measure What reviews report Average fat injected About 300 mL per breast (range 134–610 mL) in a systematic review of 35 studies including 3,757 women Average volume retained About 58% (range 44–83%); another review reported 39% to 76% Overall complication rate 27.8% of cases; fat necrosis made up 43.7% of all complications Imaging changes Fat necrosis in 9.4% and calcification in 1.2% of cases Patient satisfaction

Reported as high despite incomplete retention
If about 300 mL is injected and about 58% survives, the gain is around 175 mL — by the common rule of thumb that one cup is roughly 150–200 mL, about one cup size. Results vary with blood supply, surgical technique, recipient tissue, smoking and weight change. Risks include fat necrosis and oil cysts, calcifications visible on mammograms, bruising, swelling, infection, asymmetry, donor-site dents or numbness, and anaesthesia risks.

Promising but demanding: external expansion with fat transfer

Some clinics use a bra-like vacuum device worn for long periods each day for several weeks before fat grafting. The aim is to stretch the tissue gently and improve its blood supply, so that it accepts more fat and more of it survives. A developer-led study of 81 patients reported an average gain of 233 mL per breast at 12 months, compared with 134 mL in published series of fat transfer without expansion — though the comparison was against published series rather than a randomly assigned control group. A 2018 review of 13 studies reported graft survival between 53% and 82% at 6 to 12 months, with high satisfaction. A 2022 review concluded that external expansion improves fat survival but that complications and poor compliance are obvious, and that no standard parameters exist.

The burden is heavy: many hours of device wear a day for weeks, skin problems such as swelling, bruising, blisters and dermatitis, and rare but reported serious complications including pneumothorax. The verdict is promising, not settled — the strongest numbers come from the group that developed the technique.

Add-ons and devices

Treatments that are real — but do not enlarge the breast

Treatment What it can do What it is sold as Verdict PRP alone May support tissue repair elsewhere in the body Non-surgical breast enlargement or lift UNPROVEN for breast volume Stem-cell or SVF enrichment Experimental add-on to fat grafts Higher fat survival and bigger result UNPROVEN ; may reduce usable volume Muscle stimulators (EMS) Contract the chest muscle Breast lifting and growth DOESN’T WORK for size Radiofrequency, ultrasound, laser Mild skin tightening Firmer, bigger breasts DOESN’T WORK for size Thread lifts Reposition tissue for a limited time Instant enlargement DOESN’T WORK for size

Methods that don’t work: products you can buy

Creams and lotions act on the skin surface. They can moisturise and make the skin feel smoother, and a smoother surface can look firmer for a while. There is no plausible way for a cream to grow glandular or fatty tissue underneath, and no well-designed clinical trial shows that any cream does so. Beware of creams that contain hormones that are not listed on the label.

Herbal pills, teas and capsules usually contain plant extracts promoted as “phytoestrogens”. A medical review of bust-enhancing herbal products published in Obstetrics & Gynecology found that no clinical trials had been published, noted that several ingredients — including hops, saw palmetto, dong quai, chaste-tree berry, wild yam, fennel, black cohosh and fenugreek — are hormonally active, and concluded that their use should be discouraged because of lack of evidence of efficacy and long-term safety concerns. Hormone-active ingredients can cause temporary breast fullness and tenderness, which looks like growth — and can also disrupt periods and hormone levels.

Foods and diets (soy, flaxseed, milk, papaya and similar claims) target nothing: the amount of plant compounds in a normal diet does not enlarge the breast. Massage may relax and improve skin feel, but forceful massage can bruise and is not advisable. Suction and vacuum cups draw fluid and blood into the tissue, causing temporary swelling that fades in hours or days — the exact effect people mistake for growth — and misuse can cause bruising, broken blood vessels and skin damage. Hypnosis, subliminal audio, magnets, patches and acupuncture have no credible mechanism and no credible evidence; some patches even contain hormones or other active ingredients.

People believe these methods work because of temporary swelling, cyclical changes across the menstrual cycle, small weight gains during weeks of use, better posture or a better bra, hope and the placebo effect, and the selective sharing of success stories.

Methods to avoid: fillers, silicone and unsupervised hormones

The US FDA recommends against using dermal fillers or any injectable filler to increase breast size — these uses are not approved, and the risks of unapproved uses are not known. In a 2017 safety communication, the FDA warned that injectable silicone and products falsely marketed as approved fillers have caused serious injuries and disfigurement when used to enlarge the breasts; large-scale injections can block blood vessels in the lungs, heart or brain, causing stroke or death, and problems can appear years later. At an August 2025 advisory meeting, the FDA raised concerns that filler can migrate, form nodules or granulomas, interfere with mammograms and breast examination, and potentially affect breastfeeding — and news coverage reported that no approved method exists for removing dermal fillers. Needle-free devices are not approved for injecting fillers, no filler is approved for over-the-counter use, and people should never inject themselves. Hormone products bought online are not a cosmetic route to larger breasts and carry real risks including blood clots.

Spotting false claims

Red flags before you spend money

Claim you will see Why it is a problem A better question “Clinically proven” Often means an in-house survey or a study on something else Where is the independent, published trial, and what did it measure? “100% natural” Natural ingredients can still be hormone-active or contaminated What are the ingredients, and has anyone tested their effect on breast size? “Doctor recommended” Which doctor, and were they paid? Can I speak to a registered specialist independent of the seller? “Results in 30 days” Fast changes are usually swelling or weight change How was the result measured, and how long did it last? “FDA registered” In the US, registration of a facility is not the same as approval of a product for a use Is this product approved or cleared for this specific use? “Permanent, no risk” No breast procedure is risk-free, and few results are permanent What are the complication rates, and what happens if I am unhappy? Before-and-after photos Lighting, posture, timing and bras change the picture Are they standardised, unretouched and from real patients like me? “Limited-time offer” Pressure removes time to think Can I take the written plan home and decide later?
Before you buy: search the product name with “warning”, “recall” or “adverse events”; check the regulator in your country for approval status; look for a published, independent study; read the full ingredient list; and ask a pharmacist or doctor about interactions with your medicines or conditions.

Compared side by side

Effort, risk and reversibility

Method Effort and time Invasiveness Time to a visible change Reversible? Risk level Bras, padding, styling Minutes; ongoing None Immediate Yes None Strength training 2–3 sessions a week None Weeks to months Yes Low Fat transfer One or two procedures, then recovery Minimally invasive surgery Swelling first; final result after several months Not fully; lost fat cannot be replaced without repeating Moderate External expansion plus fat transfer Weeks of daily device wear plus a procedure Device plus surgery Months Partly Moderate Creams, massage Daily None No lasting change Yes Low Herbal or hormone pills Daily for weeks Oral No lasting change; swelling possible Effects of hormones may persist Low to moderate Suction cups Sessions of minutes to hours External Hours (swelling) Yes Low to moderate Fillers in the breast A clinic visit Injection Immediate No reliable method of removal High Liquid silicone, oils A clinic or unlicensed visit Injection Immediate No Very high The effort-to-reward trade: methods that ask little of you give little and cost little. Methods that give real volume ask for a procedure, recovery time and acceptance of risk. No method on this list gives a large, lasting gain for no effort and no risk, and any seller who says otherwise is selling hope.

What to do instead

A sensible order of steps

Match the method to your actual goal, then pick the least invasive option that can deliver it.

Look fuller in clothes

Start with a professional bra fitting, padded or push-up styles, necklines and posture. Rarely needs a specialist.

Get a firmer chest

Strength training and posture work for 8 to 12 weeks. Consider a specialist if loose skin or marked sagging remains.

A modest, lasting increase

Learn about fat transfer and ask for retention and complication data — if you have stable weight, enough donor fat and realistic goals.

A large, predictable increase

Learn about surgical options, and see a qualified plastic surgeon. Compare more than one opinion, in writing, with the full cost and a revision policy.

Different-sized breasts

A padded insert on the smaller side; have any recent change checked. Discuss fat transfer or surgery if the difference bothers you.

Under 18

Talk with a doctor and a trusted adult — breasts keep developing for years. No product, device or procedure is appropriate.

Real-life scenarios

Common situations, sensible paths

Situation A sensible path What to avoid “I have an event in three months and want to look fuller.” Get a professional bra fitting; choose a tailored outfit and neckline; consider padded or adhesive options; work on posture. Pills and quick injections: they are too slow, unproven or unsafe. “I’m slim and want a modest, natural increase.” Ask a qualified surgeon whether you have enough donor fat for fat transfer, and compare it with surgical options. Gaining weight at random, or any filler. “My breasts changed after pregnancy or weight loss.” Get re-measured for bras; train the chest and back; ask a surgeon about lift or augmentation if it bothers you. Creams promising to “restore” volume. “An ad promises two cup sizes in 30 days.” Do not buy. Check ingredients and look for independent, published trials. Any product with hormone-like or undeclared ingredients. “A clinic has offered me filler in my breasts.” Decline, and ask for the product name, its regulatory approval for breast use and what happens if you need removal; get a second opinion from a qualified surgeon. Agreeing the same day, or paying a deposit under pressure. “One breast is clearly smaller.” Use a padded insert on the smaller side; have any recent change examined; discuss fat transfer or surgery if it bothers you. Treating one side with creams, cups or devices.
Safety

Warning signs — and what to do

Sign Why it matters What to do Fever, spreading redness, pus or worsening pain after a treatment Possible infection Contact your clinician the same day. Pale, bluish, dusky or dark skin, or blisters May mean reduced blood supply Seek urgent medical care. Chest pain, breathlessness or collapse Possible blood clot or other serious event Call emergency services. Sudden vision change, severe headache or weakness after filler Possible blocked blood vessel Call emergency services. New lump or hardness weeks or months later Fat necrosis, filler reaction or something unrelated Have it examined and imaged. Nipple discharge, dimpling or skin changes Can arise for unrelated reasons See a doctor promptly.
Whatever you choose, keep up regular breast awareness and follow your national screening advice. Tell any radiographer about fat grafting, injections or implants, and keep a record of what was done, when and by whom.

Questions

Frequently asked questions

Is there any non-surgical method that makes breasts bigger permanently?

Fat transfer is the best-supported option for lasting volume, but it involves liposuction and delivers modest, variable gains. External expansion before grafting may increase them.

Do breast enlargement pills work?

A medical review found no published clinical trials of bust-enhancing herbal products and discouraged their use. There is no controlled trial showing that any pill enlarges the breasts.

Do creams and massage work?

No reliable evidence shows that they enlarge breasts. They can improve how the skin feels.

Do chest exercises make breasts bigger?

They build the muscle beneath the breast and can improve firmness and posture, but they do not increase breast tissue.

Is filler a safe shortcut?

The FDA recommends against filler for breast enlargement and has not approved any for this use. Risks include nodules, migration, imaging problems and difficulty removing the material.

Is fat transfer safe?

It is widely used, but one review reported an overall complication rate of 27.8%, with fat necrosis the most common. Discuss the risks with a qualified surgeon.

How many cup sizes can I realistically gain without surgery?

Bras and padding add about a cup visually. Fat transfer is typically around one cup per session. More usually needs implants.

Will it affect mammograms?

Fat grafting and fillers can both cause imaging changes. Always tell your radiographer.

Why do so many people say supplements worked for them?

Temporary swelling, cycle changes, small weight gains and better bras can look like growth, and success stories are shared more than failures.

What is the single best first step?

A professional bra fitting. It costs little, carries no risk, and often changes the look more than people expect.

Sources and further reading

- U.S. Food and Drug Administration — Dermal Fillers (Soft Tissue Fillers), and the 2017 safety communication on illegal use of injectable silicone for body contouring.
- U.S. Food and Drug Administration — Executive Summary: Meeting on Dermal Fillers (advisory panel, August 2025).
- Seth I, et al. — Autologous fat grafting in breast augmentation: a systematic review highlighting the need for clinical caution (35 studies, 3,757 women).
- Adipose-derived stem cell enrichment is counter-productive for the majority of women seeking primary aesthetic breast augmentation by autologous fat transfer: a systematic review. J Plast Reconstr Aesthet Surg , 2020;73(11):2025–2032.
- Khouri RK, et al. — Brava and Autologous Fat Transfer Is a Safe and Effective Breast Augmentation Alternative: Results of a 6-Year, 81-Patient, Prospective Multicenter Study. Plast Reconstr Surg , 2012.
- The Impact of Recipient Site External Expansion in Fat Grafting Surgical Outcomes. Plast Reconstr Surg Glob Open , 2018; and Research progress of external volume expansion assisted autologous fat grafting for breast reconstruction, 2022.

- “Bust enhancing” herbal products. Obstet Gynecol , 2003;101(6):1345–9; ConsumerLab.com breast enhancement supplements review (2002).
- American Society of Plastic Surgeons — breast augmentation patient information and the 2025 plastic surgery statistics. plasticsurgery.org · 2025 statistics

Browse the complete published literature

This guide cites selected sources. The PMC research library indexes every PubMed Central record for the phrase “breast enlargement” — with abstracts, citations and direct links to the original articles.

Educational information only — not medical advice

This guide does not recommend any treatment for you. Decisions about your body should be made with a qualified, registered doctor who can examine you. If you notice a new breast lump or any urgent warning sign, seek medical care promptly.

Your journey starts with information

Talk through the options with a qualified clinician

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Breast Implant Safety: Rupture, ALCL, Contracture & Monitoring

Page: /breast-implant-safety/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Safety

Breast Implant Safety

Most people who have implants are satisfied, but implants have documented risks that do not disappear with time. This page sets out what the evidence and regulators actually say.

Implant safety is not a single yes-or-no. It is a set of known, quantified risks — some common and manageable, some rare and serious — against benefits that many patients find worthwhile. Reading them honestly is the basis of informed consent.

The main risks

What can happen, and how common it is

Risk What it is What to understand Capsular contracture Tightening of the scar capsule around the implant The most common reason for revision; can cause firmness, distortion and discomfort. Textured surfaces were introduced partly to reduce it. Rupture / silent rupture Loss of implant integrity Silicone rupture may cause no symptoms, which is why screening is recommended; saline rupture is usually obvious as the breast deflates. BIA-ALCL Breast implant-associated anaplastic large cell lymphoma A rare lymphoma of the fluid or capsule, linked mainly to textured implants. It is treatable, particularly when caught early. BIA-SCC Breast implant-associated squamous cell carcinoma A very rare cancer in the capsule; regulators list it alongside ALCL as a device-associated risk. Infection Infection around the implant May require antibiotics and, in some cases, implant removal. Systemic symptoms (“BII”) Self-reported fatigue, “brain fog”, joint pain and rash Widely reported but not an established diagnosis; reviews find the correlation with implants is not established and evidence is limited. Sensation & breastfeeding changes Altered nipple or skin sensation; effects on feeding Varies with technique and individual anatomy; discussed before consent. Need for revision Further surgery at some point Reported revision rates after primary augmentation are as high as 36%.

Monitoring, not reassurance

For silicone implants the FDA recommends MRI or ultrasound at five to six years after placement and then every two to three years, because a ruptured implant can produce no symptoms. Ongoing monitoring is part of living with implants — not a sign that something has gone wrong.

What the systematic reviews show

The Cochrane Library holds independent systematic reviews and registered protocols on breast implants, wound drains, implant types and reconstruction. Systematic reviews pool many studies and sit at the top of the evidence ladder, which makes them the most reliable single guide to what is known and, equally importantly, to where the evidence is still missing.

The literature also documents past device failures and withdrawals — from PIP implants, with their increased rupture and leakage risk, to the soybean-oil-filled Trilucent implants that were withdrawn because of lipid-oxidation metabolites. These episodes are why modern regulators require long-term data and registries.

Injectable silicone and fillers

Importantly, most of the serious “breast filler” harms in the literature are not about implants at all. They concern injectable fillers and liquid silicone used to enlarge the breast off-label — products that can migrate, cause infection, granulomas and disfigurement, and interfere with mammography. The safest position on injectable fillers and silicone in the breast is the regulator position: do not use them for this purpose.

Regulator and systematic-review sources

U.S. Food and Drug Administration (FDA) — www.fda.gov/

- Breast Implants (Regulatory hub)
- Types of Breast Implants (Regulatory page)
- Risks and Complications of Breast Implants (Regulatory page)
- Things to Consider Before Getting Breast Implants (Patient decision guide)
- Breast Implant Surgery (Regulatory page)
- Breast Implants — Certain Labeling Recommendations to Improve Patient Communication (FDA guidance (2020))
- …and 10 more in the full library.

Cochrane Library (Cochrane Database of Systematic Reviews) — www.cochranelibrary.com/

- Wound drainage after plastic and reconstructive surgery of the breast (Cochrane Review)
- Different types of implants for reconstructive breast surgery (Cochrane Review)
- Implants versus autologous tissue flaps for breast reconstruction following mastectomy (Cochrane Review)
- Breast implant insertion funnels for preventing capsular contracture (Cochrane Protocol)
- Mesh for immediate implant-based prepectoral breast reconstruction following mastectomy (Cochrane Protocol)
- Acellular dermal matrices for breast reconstruction surgery (Cochrane Protocol)
- …and 1 more in the full library.

Educational information only

This page summarises published evidence and regulator guidance for general education. It cannot account for your health or circumstances and does not recommend any treatment. Discuss any symptom after a procedure promptly with a qualified clinician.

Your journey starts with information

Have questions about implant safety?

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Breast Implants: Types, Placement & Long-Term Reality

Page: /breast-implants/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Surgical pathway

Breast Implants: Types, Placement and Long-Term Reality

Breast implants are medical devices placed surgically to increase breast volume. They are the most studied enlargement method — with real complication and revision rates that every patient should understand before deciding.

Not the same as injectable silicone

Injectable silicone is a different product from a silicone gel-filled implant and must never be used as a substitute. Injectable silicone used to enlarge the breasts can cause serious, potentially permanent harm.

An implant is a device — a shell filled with saline (sterile salt water) or silicone gel — placed behind the breast tissue or behind the pectoral muscle. Implant surgery is the best-documented route to a larger, predictable breast size, but it is surgery: it carries risks, it is not permanent, and follow-up over the years is part of the decision.

Implant types

Filling, surface and shape

Choice Options Trade-offs to understand Filling Saline (salt water) or silicone gel Silicone gel tends to feel more like natural breast tissue and does not collapse if it leaks; it requires monitoring for silent rupture. Saline collapses and is absorbed if the shell leaks. Regulators approve saline from age 18 and silicone from age 22 in the US. Surface Smooth or textured Smooth implants tend to feel softer and move naturally but may ripple; textured implants are less likely to move but are where implant-associated ALCL occurs most often. Shape Round or form-stable (“gummy bear”) Round gives a fuller look with less concern about rotation; shaped implants give more controlled projection but can look unusual if they rotate and need a longer incision. Profile Low to high projection Profile determines how far the implant projects from the chest wall; it must match the width of your chest to look proportionate.

Placement and incisions

The implant can sit under the pectoral muscle (submuscular) or behind the breast tissue over the muscle (subglandular). Access is usually through the inframammary fold (under the breast), around the areola , or in the armpit (transaxillary). The route affects scarring and, to some extent, how the implant behaves.

There is no single best combination. Nava and colleagues reviewed the evidence and found no high-level evidence identifying one ideal technique or implant; complication reduction follows established surgical principles rather than device fashion.

Longevity, monitoring and revision

Breast implants are not lifetime devices . There is no fixed lifespan: some remain stable for many years, but

additional operations are common over a lifetime of implant use. For silicone implants, the US FDA recommends MRI or ultrasound screening five to six years after placement and then every two to three years, because a rupture can be silent.

Revision rates after primary augmentation have been reported as high as 36% , most commonly for capsular contracture, implant malposition and ptosis of the ageing breast. None of this means implants are unsafe — most people are satisfied — but it should shape expectations.

Related pages

Go deeper

[Image: Bold abstract breast contour lines on a burgundy background]

Breast implant safety

Rupture, capsular contracture, ALCL, screening and what the regulators say about monitoring.

[Image: Abstract breast-focused medical artwork in burgundy, rose and champagne gold]

Breast augmentation

How the operation is planned and performed, recovery, and what it can and cannot achieve.

[Image: Illustrative breast anatomy artwork for patient education]

Choosing a surgeon

How to check credentials, registrations and the questions worth asking before committing.

Where this page is grounded

NCBI Bookshelf (StatPearls Publishing) — www.ncbi.nlm.nih.gov/books/

- Continuing Education Activity (StatPearls review section)
- Introduction (StatPearls review section)
- Anatomy and Physiology (StatPearls review section)
- Indications (StatPearls review section)
- Contraindications (StatPearls review section)
- Equipment (StatPearls review section)
- …and 7 more in the full library.

American Society of Plastic Surgeons (ASPS) — www.plasticsurgery.org/

- Breast Augmentation (Augmentation Mammaplasty) (Procedure overview)
- Breast Augmentation Candidates (Patient guidance)
- Breast Augmentation Consultation (Patient guidance)
- Questions to Ask Your Plastic Surgeon (Question checklist)
- Breast Augmentation Cost (Cost guidance)
- Types of Breast Implants (Device overview)
- …and 7 more in the full library.

Educational information only

Implant selection, technique and suitability are individual decisions that require examination and assessment by an appropriately credentialed surgeon. Nothing here is a recommendation for any individual.

Your journey starts with information

Discuss implants with a qualified surgeon

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Choosing a Surgeon: Credentials, Questions and Red Flags

Page: /choosing-a-surgeon/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Decisions

Choosing a Surgeon

The single biggest determinant of a safe, well-judged result is the clinician you choose. Credentials can be checked, and the questions that expose a weak consultation can be asked.

Advertising is not a credential. Anyone can describe themselves as a “cosmetic surgeon”, and clinics may use the word “expert” freely. What can be verified is registration with a professional regulator, recognised surgical training, and membership of a relevant professional body.

Verification

How to check a surgeon before you commit

These steps are cheap, quick and entirely reasonable to insist on.

[Image: Stylised breast contour illustration with dramatic burgundy light]

Confirm registration

Find the surgeon on your national medical council or regulator and confirm the registration is current and unrestricted.

Confirm surgical training

A surgeon performing breast surgery should hold recognised training in plastic or reconstructive surgery, not merely a short cosmetic course.

Check scope of practice

The clinician should operate within their qualifications; ask directly who will perform each part of the procedure.

Check the operating setting

Ask where the procedure is performed, who provides anaesthesia, and what arrangements exist for emergencies.

Ask for the plan in writing

A reputable surgeon provides a written plan covering technique, risks, costs and revision policy — so you can think it over.

Questions

What to ask at the consultation

Question What a good answer looks like How many of these procedures do you personally perform each year? A specific number and a willingness to discuss outcomes, not evasion. What are your complication and revision rates? Honest acknowledgment that complications occur, with figures or a range, not a claim of zero. What would you do if I were unhappy with the result? A clear revision policy and costs, stated in advance. Which

options are you not offering me, and why? A balanced comparison — including options that are less profitable or out of scope. How do you handle a problem after surgery? A named route for concerns, including out-of-hours arrangements. Can I take the written plan home before deciding? Yes, without pressure. A cooling-off period is normal and expected.

Red flags that should end a consultation

- Pressure to decide or pay a deposit the same day, or a “limited-time” discount.
- Guaranteed results, guaranteed symmetry, or claims of “no risk”.
- Vague or evasive answers about who performs the surgery and where.
- Reluctance to provide a written plan, quote or revision policy.
- Pushing an unapproved injectable, or refusing to name the exact product being offered.
- No discussion of alternatives, contraindications or what happens if things go wrong.

Why a cooling-off period matters

Cosmetic surgery decisions are permanent enough that reflection is part of good practice. Professional bodies such as the UK’s BAPRAS describe a consultation and cooling-off period before consent precisely because a rushed decision is a poor decision. If a clinic discourages taking time, that in itself is information.

Professional-society guidance

bapras

American Society of Plastic Surgeons (ASPS) — www.plasticsurgery.org/

- Breast Augmentation (Augmentation Mammaplasty) (Procedure overview)
- Breast Augmentation Candidates (Patient guidance)
- Breast Augmentation Consultation (Patient guidance)
- Questions to Ask Your Plastic Surgeon (Question checklist)
- Breast Augmentation Cost (Cost guidance)
- Types of Breast Implants (Device overview)
- …and 7 more in the full library.

Your journey starts with information

Prepare your questions in advance

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Contact Us | Quantum Health

Page: /contact/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Contact

Contact Us

This website is an educational resource, not a clinic. Use this page for corrections, source suggestions or general questions about the content.

This website does not provide treatment, consultations or diagnoses, and no clinician is available through it. Please read the notes below before writing to us.

What to contact us about

Appropriate and inappropriate enquiries

Content corrections & sources

Spotted an error, a broken link or a source we should include? We welcome evidence-based corrections and suggestions.

General questions about the content

Questions about how a page explains a concept, or where a cited source comes from, are welcome.

Medical advice — not here

We cannot give personal medical advice, diagnose a condition or recommend a treatment. Please see a qualified, registered clinician.

Urgent symptoms — seek care now

If you have fever, spreading redness, a new lump, chest pain or any urgent symptom after a procedure, contact a clinician or emergency services immediately.

Message form

Send a general message

This form collects only what is needed to reply. Please do not send detailed medical history, symptoms, test results or images. We cannot respond to individual medical questions.

- We reply by email only.
- We cannot provide medical advice or diagnosis.
- Please do not include personal medical details.
- Messages are handled confidentially.

[Image: Illustrative breast anatomy artwork for patient education]

This is not a clinic

No doctor, clinic, medical team or physical address is presented on this website because none has been verified. Submitting a message does not create a doctor–patient relationship and does not commit you to any treatment.

Prefer to email us directly?

Write to enquiries@nonsurgicalbreastenlargement.in . This is the only contact channel published on this website.

Cookie Policy | Quantum Health

Page: /cookie-policy/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Policy

Cookie Policy

How cookies and similar technologies are used, and how you can manage consent.

What cookies are

Cookies are small files stored by your browser. This website aims to use as few as possible and to load any non-essential cookies only after consent.

Categories

Essential: required for the website to function. Analytics: used to understand site usage — loaded only after consent. Marketing: used only if applicable and only after separate consent. [To be completed once tooling is confirmed.]

Managing consent

[To be completed: consent-management mechanism and instructions for withdrawing consent at any time.]

Breast Enhancement FAQs | Breast Enlargement Guide

Page: /faqs/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Patient information

Frequently Asked Questions

Straightforward answers to the questions patients most often ask. Every individual situation still requires assessment by an appropriately qualified clinician.

Common questions

Breast enhancement FAQs

Can breast size be increased without surgery?

Some treatments are marketed for this purpose, but the evidence and regulatory status vary. Many non-surgical approaches have not been shown to reliably increase breast volume. A professional assessment is the appropriate way to establish what is realistic and which options, if any, are appropriate.

Are dermal fillers safe for breast enlargement?

Dermal fillers are not approved for breast enlargement, and regulators advise against using them for this purpose because of potentially serious complications. A filler approved for facial use should not be assumed suitable for injection into the breasts.

Is PRP a proven breast enlargement treatment?

PRP’s effects depend on the specific indication and the evidence available. It should not be marketed as a proven method of increasing breast size without robust evidence supporting that particular use.

What is the difference between silicone implants and injectable silicone?

They are entirely different. Injectable silicone is a different product and should not be used as a substitute for a breast implant. Injectable silicone used for breast enhancement can cause serious, potentially permanent harm.

Does fat transfer produce permanent results?

Some transferred fat can survive long term, but a proportion may be reabsorbed. The final result varies between individuals, and additional procedures may be considered.

How long does breast augmentation recovery take?

Recovery varies according to the procedure, surgical technique, individual healing and the clinician’s advice. Your surgeon will give you a personalised return-to-activity timeline.

How do I know which procedure is right for me?

The appropriate option depends on your health, anatomy, expectations, risk tolerance and the evidence for the intended treatment. The decision should follow an assessment by an appropriately qualified clinician.

Your journey starts with information

Still have questions? Arrange a consultation

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Glossary: Breast Enlargement and Implant Terms

Page: /glossary/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Reference

Glossary of Breast Enlargement Terms

Plain-language definitions of the terms you will meet on this site and in a consultation.

Terms are arranged alphabetically. Definitions are deliberately short and non-promotional — enough to follow a consultation or a research abstract, not a substitute for clinical advice.

autologous fat grafting

The transfer of a patient's own (autologous) fat from one part of the body to another; the scientific name for fat transfer.

BIA-ALCL

Breast implant-associated anaplastic large cell lymphoma — a rare lymphoma of the fluid or capsule around an implant, linked mainly to textured implants.

BIA-SCC

Breast implant-associated squamous cell carcinoma — a very rare cancer in the capsule around an implant.

biostimulator

A broad category of injectable products intended to stimulate a tissue response such as collagen production, for selected approved indications.

breast augmentation

Surgery to increase breast size or change shape, using implants or, in selected cases, the patient's own fat.

breast implant

A medical device placed surgically to increase breast volume or restore shape, filled with saline (salt water) or silicone gel.

breast implant illness

A term used for self-reported systemic symptoms such as fatigue, "brain fog", joint pain and rash attributed to implants; not an established diagnosis.

breast reduction

Surgery that removes breast tissue, fat and skin to reduce breast size and weight, often to relieve physical symptoms.

capsular contracture

Tightening and hardening of the scar tissue capsule that naturally forms around an implant; a common reason for revision surgery.

capsulectomy

Surgical removal of the scar capsule around a breast implant, sometimes performed at the same time as implant exchange or removal.

cohesive gel implant

A silicone implant whose gel holds together, so it tends to stay in shape even if the shell is cut; often called form-stable or "gummy bear".

dermal filler

An injectable product used for certain approved aesthetic indications; not approved for breast enlargement.

explantation

Surgical removal of breast implants, with or without replacement.

external volume expansion

Pre-stretching the breast with a bra-like vacuum device worn for hours a day for weeks before fat transfer, aiming to improve fat survival.

fat necrosis

Death of transferred fat cells that fail to establish a blood supply; may cause lumps, oil cysts or calcification visible on imaging.

fat transfer

Enlargement or reshaping using the patient's own fat, removed by liposuction, processed and injected into the breast. Also called lipofilling or autologous fat grafting.

haematoma

A collection of blood (bruise) that can form after surgery and may require drainage.

hyaluronic acid filler

A gel filler based on hyaluronic acid, used for approved aesthetic indications; its suitability for the breast is not established.

implant rupture

Loss of integrity of a breast implant shell. A silicone rupture can be "silent", producing no symptoms, which is why screening is advised.

inframammary fold

The natural crease under the breast; a common and well-hidden incision site for implant placement.

lipofilling

Another name for fat transfer: injecting the patient's own fat, usually after liposuction, to add volume or refine shape.

liquid silicone

Injectable silicone that is not an implant and must never be used to enlarge the breast; linked to disfigurement, embolism and death.

mammography

X-ray imaging of the breast used for screening and diagnosis; implants, fat transfer and fillers can change its appearance.

mastopexy

A breast lift: reshaping and raising sagging breasts without deliberately adding volume.

MRI screening

Magnetic resonance imaging used to check silicone implants for silent rupture, recommended by the FDA at intervals after placement.

oil cyst

A pocket of liquefied fat that can form after fat necrosis following fat transfer; usually benign but may require assessment.

periareolar

An approach or incision around the edge of the areola.

poly-L-lactic acid

A biodegradable collagen-stimulating injectable (for example Sculptra), licensed for specific facial indications, not for breast enlargement.

PRP

Platelet-rich plasma: a preparation made by concentrating platelets from the patient's own blood, used in various regenerative procedures.

ptosis

Sagging or drooping of the breast; its degree helps determine whether a lift is appropriate.

revision surgery

An operation to adjust, exchange, reposition or remove breast implants after a previous procedure.

saline implant

A breast implant filled with sterile salt water; if the shell leaks, the implant collapses and the fluid is absorbed or expelled.

seroma

A collection of fluid that can form after surgery or around an implant.

silicone implant

A breast implant filled with silicone gel, which tends to feel more like natural breast tissue and does not collapse if the shell leaks.

subglandular

Placing a breast implant behind the breast tissue but in front of the pectoral muscle.

submuscular

Placing a breast implant behind the pectoral muscle.

transaxillary

An approach to breast augmentation through an incision in the armpit (axilla).

A note on terminology

Some terms — such as “regenerative” or “non-surgical” — are used in marketing far more loosely than their technical meaning allows. Where that happens, this site explains the gap rather than repeating the claim.

Your journey starts with information

Follow a term through to the evidence

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Breast Enlargement: Non-Surgical & Surgical Options Explained

Page: /

Independent Patient Education

A More Informed Approach to Breast Enhancement

Confidence begins with clear, honest information.

Understand surgical and non-surgical breast enlargement options through clear explanations, published evidence and realistic limitations — before you speak to any clinic.

This website provides general education only. Individual suitability, outcomes and risks vary and require assessment by a qualified clinician.

[Image: Cinematic abstract breast anatomy artwork with bold rose contours and champagne highlights]

Our approach

Clear information. Honest limits. No promotion.

Every person considering breast enhancement has different anatomy, breast tissue, aesthetic preferences and expectations. This website exists to explain the options before you discuss them with any clinic or surgeon.

We aim to make complex treatment choices easier to understand by presenting the potential benefits, limitations, recovery considerations and risks of each approach — so you can make decisions grounded in accurate information rather than promotion.

Whether you are exploring breast augmentation surgery, fat transfer or a non-surgical treatment, our focus is on transparent, evidence-based information and realistic expectations.

Read about this website

[Image: Breast anatomy-inspired editorial illustration with layered tissue contours]

Clarity before any decision

Our principles

Four commitments behind every page

Reader-first

Written for people weighing real decisions, not to sell any treatment.

Safety-first

Contraindications, alternatives and potential complications are stated plainly.

Evidence-informed

Explanations grounded in published research and regulator guidance, with sources linked.

No promotion

No clinic, product or practitioner is advertised or recommended here.

Treatment pathways

Explore your options by category

Two broad pathways, each with different intended outcomes, evidence and risks.

[Image: Breast-medical editorial artwork with bold rose contours and gold highlights]

Injectables · regenerative · skin quality

Non-surgical options

Selected injectable, regenerative and skin-quality approaches, considered with attention to evidence, suitability and approved indications. Not every non-surgical treatment is shown to increase breast volume.

[Image: Stylised breast contour illustration with dramatic burgundy light]

Implants · autologous fat transfer

Surgical options

Understand breast implants, silicone gel-filled implants and autologous fat transfer, including recovery and long-term considerations.

An important clinical distinction

Injectable fillers, threads, PRP and biostimulators should not be presented as interchangeable alternatives to breast augmentation surgery. Their evidence, indications and ability to increase breast volume differ substantially, and a procedure being described as “non-surgical” does not by itself make it safe.

Treatments

A closer look at individual options

[Image: Abstract breast tissue contours in a bold rose-pink and champagne-gold medical-editorial style]

Breast dermal fillers

Injectable products for specific approved aesthetic indications — and why breast use raises safety questions.

[Image: Breast anatomy-inspired medical illustration with luminous rose tissue layers and gold details]

PRP

How platelet-rich plasma is prepared, what it is intended to do, and how the evidence is assessed.

[Image: Abstract breast contour illustration with fine rose and gold lines]

Thread-based procedures

PDO and mono threads: intended mechanical effects, limitations and product-specific indications.

[Image: Abstract breast-medical artwork with layered tissue forms and rose-gold highlights]

Sculptra & biostimulators

Poly-L-lactic acid and collagen-stimulating products — approved uses versus breast-use claims.

[Image: Abstract breast contour and implant-inspired medical artwork in burgundy and rose]

Breast implants

Medical devices with an outer shell and gel filling, and their long-term monitoring needs.

[Image: Stylised breast tissue illustration with bold gold and rose details]

Fat transfer

Using your own fat for a modest, individualised increase in volume, and its retention variability.

[Image: Abstract breast-focused medical artwork in burgundy, rose and champagne gold]

Breast augmentation

Surgical increase of breast volume using implants or, in selected cases, your own fat.

[Image: Bold abstract breast contour lines on a burgundy background]

Compare all options

A side-by-side view of intended effect, primary consideration and important limitation.

How to use this website

A clear, five-step pathway to an informed decision

Work through the information at your own pace. No decision about your body should ever be rushed by advertising.

[Image: true]

Learn what each option is

Start with the non-surgical and surgical sections to understand intended effects, mechanisms and limitations.

Check what the evidence shows

Read the evidence guide and the research libraries to see what is proven, what is unproven and what to avoid.

Understand the risks

Review candidacy, contraindications, possible complications and the warning signs that need medical attention.

Compare fairly

Use the side-by-side comparison to weigh intended effect, evidence quality, recovery and reversibility.

Talk to a qualified professional

Take your written questions to a registered, appropriately qualified clinician and compare more than one opinion before deciding.

Education and insights

Understand the differences before you decide

Evidence-led articles on treatment options, safety and the questions worth asking.

[Image: Abstract breast anatomy artwork for patient education]

Treatment guides

Surgical vs non-surgical breast enhancement

Understand the differences in intended outcome, recovery, evidence and risk.

[Image: Breast-medical editorial illustration with gold and rose tissue contours]

Patient resources

Questions to ask before treatment

A practical guide to credentials, products, informed consent and aftercare.

[Image: Breast anatomy-inspired abstract contours with luminous rose highlights]

Evidence & research

PRP and the evidence

The difference between researching a treatment and establishing its safety and effectiveness.

Research & sources

Go straight to the published sources

This website is built from primary sources. Start at the research hub, or go directly to a topic library — every page links to the original regulator guidance, public-health information, systematic reviews or peer-reviewed record behind it.

[Image: Breast anatomy-inspired abstract contours with luminous rose highlights]

Start here

Research hub

An index of every research and reference library on this website, with a short explanation of what each one contains and how to read it.

[Image: Abstract breast anatomy artwork with soft rose tissue forms]

PubMed

PubMed research

Peer-reviewed breast enlargement studies indexed in PubMed — fat transfer, implants, external expansion, fillers and global trends, grouped by topic with a link to every record.

[Image: Breast-focused medical editorial artwork with bold gold and rose arcs]

PubMed Central

PMC research library

Every PubMed Central record for “breast enlargement” — searchable, with abstracts and links to the original articles.

[Image: Breast-focused medical editorial artwork with bold gold and rose arcs]

Cochrane Library

Cochrane evidence library

Every Cochrane systematic review and registered protocol on breast implants, wound drains, implant types and reconstruction, with key findings and evidence gaps.

[Image: Breast-focused medical editorial artwork with bold gold and rose arcs]

U.S. FDA

FDA information library

US regulator pages on breast implants, dermal fillers and injectable-silicone warnings, summarised and linked.

[Image: Abstract breast anatomy contours in rose against a dark burgundy ground]

NHS (UK)

NHS information library

UK public-health patient information on breast enlargement, costs, complications and choosing a practitioner.

[Image: Abstract breast anatomy contours in rose against a dark burgundy ground]

ASPS (US)

ASPS library

American Society of Plastic Surgeons patient information on breast augmentation: candidacy, cost, implant types, safety, recovery, results and choosing a surgeon.

[Image: Breast anatomy-inspired abstract contours with luminous rose highlights]

Bibliography

All references

A single consolidated bibliography of every source cited across this website, grouped by publisher and linked to the original.

Your journey starts with information

Keep exploring the evidence

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Medical Disclaimer | Quantum Health

Page: /medical-disclaimer/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Policy

Medical Disclaimer

Important limitations on the medical information published on this website.

Website content is provided for general educational purposes and is not a substitute for professional medical advice. This website is not a clinic and does not provide treatment, consultations or diagnoses.

Treatment availability, suitability, risks and results vary. Reference to any product, brand, technique or regulatory position does not confirm its approval, availability or suitability in your jurisdiction. Regulatory and evidence positions change over time; always check the original source.

Always seek the advice of an appropriately qualified medical professional with any questions about a medical condition or treatment. If you notice a new breast lump or any urgent symptom, seek medical care promptly.

Biostimulators: Collagen-Stimulating Treatments | Breast Enlargement Guide

Page: /non-surgical/biostimulators/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Non-surgical · Biostimulators

Biostimulators: Collagen-Stimulating Treatments Explained

Biostimulators are a broad category of products intended to stimulate a tissue response, such as collagen production, for selected indications. Their ingredients, mechanisms, authorised uses and safety profiles vary considerably.

What this page should explain

Points to clarify

Product-specific indications

The authorised indication for each individual product.

Onset & limitations

Expected onset and the limitations of any authorised effect.

Possible complications

Potential nodules, inflammation, infection and other complications.

Alternatives

Alternatives for patients whose primary goal is increased breast volume.

Because this category is broad, it is important to identify products accurately and to distinguish their approved uses from experimental or unsupported claims. This page is intended as a starting point for a specific conversation at consultation.

Your journey starts with information

Discuss biostimulators with a qualified clinician

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Breast Thread Procedures: Uses & Limitations | Breast Enlargement Guide

Page: /non-surgical/breast-threads/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Non-surgical · PDO threads

Breast Thread Procedures: Uses & Limitations

Thread procedures use medical sutures placed in tissue for specific intended effects. Different thread materials, designs and placement techniques have different indications and limitations.

Understanding thread-based procedures

Thread-based procedures are not, on current evidence, a predictable alternative to breast surgery. Threads should not be presented as established substitutes for breast augmentation.

Potential risks include infection, discomfort, visible or palpable threads, asymmetry, thread migration or extrusion, and the need for corrective treatment. Suitability depends on the precise product, intended use, clinician competence and applicable regulations.

What this page should cover

Points to review at consultation

Product-specific indications

Thread material and the indications specific to each product.

Intended outcome & limits

What the procedure is intended to do, and what it cannot reasonably achieve.

Alternatives & requirements

Available alternatives and the consultation requirements before any procedure.

Questions

Thread FAQ

Are threads an alternative to breast surgery?

Threads are not established substitutes for breast augmentation. Their evidence, indications and ability to increase breast volume differ substantially from surgery.

What are the risks of thread procedures?

Risks can include infection, discomfort, visible or palpable threads, asymmetry, thread migration or extrusion, and the possibility that corrective treatment is needed.

Your journey starts with information

Discuss thread options and their limitations

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Breast Fillers: Safety, Evidence & Alternatives | Breast Enlargement Guide

Page: /non-surgical/dermal-fillers/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Non-surgical · Injectable fillers

Breast Dermal Fillers: Safety, Evidence & Alternatives

Dermal fillers are injectable products used for certain approved aesthetic indications. Their use for breast enlargement is a different matter and is not established, approved or routine.

Regulatory position

The FDA advises against using dermal fillers or other injectable fillers to increase breast size. Serious complications from inappropriate body injections can include infection, persistent pain, tissue damage, scarring, deformity and potentially life-threatening events.

What dermal fillers are

Dermal fillers are injectable products used for certain approved aesthetic indications, depending on the product and the jurisdiction. Different products have different compositions, behaviours and licensed uses.

Why breast use is different

An approval for facial use does not imply suitability for injection into the breasts. The breast is a different anatomical site with different considerations, and the consequences of complications can be more difficult to manage. Because of this, breast filler injections should not be advertised or presented as a routine breast-enlargement service.

What this page should help you understand

Safety and decision points

Potential complications

The difficulty of managing some injected materials, and why complications can be serious or persistent.

Evidence-based alternatives

Options for people seeking increased breast volume that are supported by clearer evidence.

Independent advice

The importance of independent medical advice before considering any advertised treatment.

Questions

Dermal filler FAQs

Are breast fillers approved?

Dermal fillers are not approved for breast enlargement. Regulators advise against using them for this purpose because of potentially serious complications.

What alternatives exist for added volume?

Alternatives with clearer evidence for increasing breast volume include surgical options such as implants or, in selected cases, autologous fat transfer. Suitability depends on individual factors and should be assessed by an appropriately qualified clinician.

Your journey starts with information

Prefer an evidence-based conversation?

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Non-Surgical Breast Enhancement: Options & Safety | Breast Enlargement Guide

Page: /non-surgical/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Non-surgical treatment information

Non-Surgical Breast Enhancement: Options, Evidence & Safety

Non-surgical treatments differ widely in their intended effects, evidence and regulatory status. This section explains those differences clearly, without advertising claims.

Treatments marketed as “non-surgical breast enlargement” are not a single category. They include injectable fillers, platelet-rich plasma (PRP), thread-based procedures and collagen-stimulating biostimulators. Each has a different intended effect, a different evidence base and a different regulatory position.

Crucially, being non-surgical does not automatically mean a treatment is safe, and not every treatment marketed for this purpose has been demonstrated to reliably increase breast volume. This section is written to help you understand those distinctions before any clinical consultation.

The most important point

Dermal fillers are not approved for breast enlargement, and the FDA advises against using fillers or other injectable fillers to increase breast size because of potentially serious complications. Regenerative and biostimulatory products require careful scrutiny of their evidence and regulatory status before any claim of benefit is accepted.

Options covered

Non-surgical treatments explained individually

[Image: Abstract breast tissue contours in a bold rose-pink and champagne-gold medical-editorial style]

Breast dermal fillers

Injectable products for specific approved aesthetic indications, and the serious safety questions around breast use.

[Image: Breast anatomy-inspired medical illustration with luminous rose tissue layers and gold details]

PRP (platelet-rich plasma)

Prepared from your own blood to concentrate platelets. What it is intended to do, and how evidence is assessed.

[Image: Abstract breast contour illustration with fine rose and gold lines]

PDO threads

Medical sutures placed in tissue for specific intended effects, with product-specific indications and limitations.

[Image: Stylised breast tissue contours in a bold burgundy, rose and gold palette]

Mono threads

Ultra-fine threads that differ in design and intended mechanical effect from cog threads.

[Image: Abstract breast-medical artwork with layered tissue forms and rose-gold highlights]

Sculptra & PLLA

Poly-L-lactic acid injectables for certain specified aesthetic indications, subject to product labelling and local authorisation.

[Image: Breast anatomy-inspired editorial illustration with luminous rose and champagne contours]

Biostimulators

A broad category intended to stimulate a tissue response such as collagen production, for selected indications.

[Image: Abstract breast anatomy artwork with soft rose tissue forms]

Stem cells & regenerative medicine

The subject of ongoing research. “Regenerative” does not establish that a product is effective or approved.

[Image: Breast-medical editorial artwork with bold rose contours and gold highlights]

Home care & skin quality

Supportive skin-quality measures discussed at consultation where appropriate.

[Image: Abstract breast-medical illustration with rose and gold contour lines]

What works: the evidence verdict

A scorecard of more than 20 popular methods — bras and fat transfer to creams, cups and fillers — with the evidence, the risks and the red flags behind each.

Assessing any non-surgical claim

Six questions worth asking

Any advertised non-surgical breast treatment should withstand these questions before you consider it.

[Image: Illustrative breast anatomy artwork for patient education]

What is the exact product and intended use?

Ask for the precise product name, its approved indications and whether breast use is authorised in your jurisdiction.

Is it approved for this purpose?

Facial approval does not imply approval for injection into the breasts. Products and permitted uses vary by country.

What does the evidence actually show?

Distinguish marketing claims from peer-reviewed evidence for the specific procedure being offered.

Who is performing it, and with what competence?

Suitability depends on the precise product, intended use, clinician competence and applicable regulation.

What are the risks and possible complications?

Ask about infection, pain, tissue damage, nodules, asymmetry and how complications would be managed.

What are the alternatives?

Consider evidence-based alternatives for increasing breast volume, including surgical options where relevant.

At a glance

Primary consideration and key limitation

Option Primary consideration Important limitation Dermal fillers Product-specific approved indications Not FDA-approved for breast enlargement PRP Evidence for the particular intended use Should not be promoted as proven breast enlargement Threads & mono threads Product-specific aesthetic indications Not established substitutes for breast augmentation Sculptra / biostimulators Product-specific authorised uses Do not assume suitability for breast-volume enhancement Stem-cell approaches Evidence and regulatory status of the exact product Many marketed regenerative uses remain unapproved or investigational
This table covers non-surgical options only. For a side-by-side view including surgical routes (implants and fat transfer), see the full treatment comparison . It should not be read to suggest that a non-surgical procedure is automatically safer than surgery.

Questions

Non-surgical treatment FAQs

What is the safest non-surgical way to make breasts look bigger?

A properly fitted bra, padding, clothing choices and posture change the look immediately and carry no medical risk — but they change appearance, not breast tissue. For a method-by-method verdict on every popular approach, read the evidence guide .

Does “regenerative” mean a treatment is effective?

No. The term describes an area of ongoing research, not proof of effectiveness, approval or suitability for breast enhancement. Products and permitted uses vary by jurisdiction.

Where can I find answers about fillers, PRP, fat transfer and surgery?

Each treatment page on this website answers its own specific questions, and the most common cross-cutting questions are consolidated on the main FAQs page , with links into the detailed guides.

Your journey starts with information

Understand the evidence before you choose a non-surgical option

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Mono Threads: Uses and Limitations | Breast Enlargement Guide

Page: /non-surgical/mono-threads/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Non-surgical · Mono threads

Mono Threads: Design, Effect & Limitations

Mono threads differ from cog threads in design and intended mechanical effect. These differences matter when assessing suitability and realistic outcomes.

Mono threads are fine threads used in aesthetics for particular intended effects. They should not be claimed to reliably enlarge the breast without suitable evidence. As with all thread procedures, the intended effect is defined by the specific product and technique.

What this page should cover

Review at consultation

Thread design & indications

How mono threads differ in design and intended mechanical effect.

Outcome & limitations

The intended outcome and the limits of what the procedure can achieve.

Alternatives

Available alternatives, including options with clearer evidence for added volume.

Your journey starts with information

Discuss whether mono threads suit your goals

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

PRP and Breast Enhancement: Evidence Explained | Breast Enlargement Guide

Page: /non-surgical/prp/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Non-surgical · Regenerative

PRP and Breast Enhancement: What the Evidence Says

Platelet-rich plasma is prepared from your own blood to concentrate platelets. Its effects depend entirely on the specific indication and the evidence available for it.

How PRP is prepared

PRP is prepared by taking a sample of the patient's own blood and processing it to concentrate the platelets. The resulting preparation is intended for specific purposes; those purposes are not interchangeable, and a preparation intended for one indication is not automatically appropriate for another.

PRP and breast enhancement

Claims about PRP and breast enhancement should be assessed against appropriate clinical evidence for the precise procedure being offered. The evidence for a particular use should be established before that use is described as effective.

What this page should explain

Key points

How PRP is prepared

What it is intended to do, and why preparation and indication matter.

Procedure-related risks

Possible pain, bruising, bleeding, infection and other procedure-related risks.

Clinician review

Why suitability and expected outcomes require review by a qualified clinician, with reference to local regulatory requirements.

Questions

PRP FAQs

Is PRP proven for breast enlargement?

PRP should not be marketed as a proven method of increasing breast size without robust evidence supporting that particular use. Effects depend on the specific indication and the available evidence.

Is PRP safe because it uses my own blood?

Using your own blood reduces some risks but does not remove them. Any procedure carries procedure-related risks such as pain, bruising, bleeding or infection, and suitability still requires clinical review.

Your journey starts with information

Ask what the evidence supports for your situation

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Stem Cells and Regenerative Medicine | Breast Enlargement Guide

Page: /non-surgical/regenerative-medicine/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Non-surgical · Regenerative

Stem Cells & Regenerative Medicine: An Honest Overview

Stem cells and other regenerative products are the subject of ongoing research. The term “regenerative” does not establish that a product is effective, approved or appropriate for breast enhancement.

Research versus established care

Many marketed regenerative uses remain unapproved or investigational. Products marketed as regenerative can carry risks including infection and other serious complications, and permitted uses vary by jurisdiction.

Content sections

What to understand

Meaning of “regenerative”

What the term does and does not establish about a product.

Research vs. care

The difference between an investigational approach and established clinical practice.

Sourcing & regulation

Product sourcing, processing, regulatory status, uncertainties and informed consent.

Alternatives & referral

Evidence-based alternatives and specialist referral where appropriate.

Your journey starts with information

Get a balanced view of regenerative claims

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Sculptra and PLLA: Uses, Safety & Evidence | Breast Enlargement Guide

Page: /non-surgical/sculptra-plla/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Non-surgical · PLLA

Sculptra and PLLA: Uses, Safety & Evidence

Sculptra is a poly-L-lactic acid-based injectable used for certain specified aesthetic indications, subject to product labelling and local authorisation. It should not be described as a proven breast-enlargement product.

Understanding Sculptra means understanding its intended approved uses, the difference between collagen-stimulation claims and actual breast augmentation, and the importance of respecting product-specific contraindications and instructions.

Patient education topics

What to ask about

PLLA vs. HA fillers

How PLLA-based products differ from traditional hyaluronic acid fillers.

Intended treatment area

Why the intended treatment area and approved indications matter.

Possible complications

Potential nodules, inflammatory reactions, infection and other complications.

Breast-use caution

Why breast use should not be promoted without appropriate evidence and authorisation.

Your journey starts with information

Ask about Sculptra’s approved uses

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Privacy Policy | Quantum Health

Page: /privacy-policy/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Policy

Privacy Policy

How personal information submitted through this website is handled.

Who we are

This website is an independent, educational publication about breast enlargement. It is not a clinic and does not provide medical services. This policy explains how the limited personal information collected through this website is handled.

Information we collect

If you use the contact form, we collect the information you choose to provide — such as your name, email address and message. Please do not submit detailed medical information, symptoms, test results or images; the form is not designed to receive them and we cannot act on them.

How we use it

Contact information is used only to read and, where appropriate, reply to your message. It is not sold, and it is not used for marketing. [To be completed: retention periods, legal basis, and any third-party processors used to deliver the form, as applicable to the publisher.]

Your choices

You may request access to, correction of, or deletion of the personal information you have submitted. [To be completed with the publisher’s contact point and jurisdiction-specific rights.]

Cookies and analytics

Where analytics, advertising pixels or embedded third-party media are used, they are activated only after the required consent is obtained. [To be completed once the website’s tooling is confirmed.]

References: Sources Cited Across This Website

Page: /references/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Bibliography

References and Sources

Every source this website draws on, grouped by publisher and linked to the original.

This is the consolidated bibliography behind the site. Each group below is a published collection of documents captured from a recognised regulator, public-health body, professional society or peer-reviewed source. Links open the original.

For the complete, searchable PubMed Central record set for “breast enlargement” (thousands of citations with abstracts), see the PMC research library . For a curated, topic-grouped summary of the studies most relevant to enlargement, see the PubMed research page .

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- How can I prepare for a breast augmentation? — Procedure page section · November 2025
- How soon will I recover? — Procedure page section · November 2025
- What complications can happen? — Procedure page section · November 2025
- When should I see my doctor? — Procedure page section · November 2025
- Are there alternatives to breast augmentation? — Procedure page section · November 2025
- Resources and support — Procedure page section · November 2025
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- Fat Transfer Breast Augmentation: Procedure, Results & Risks — Treatment overview · 2022-08-18

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- Breast Augmentation (Augmentation Mammaplasty) — Procedure overview
- Breast Augmentation Candidates — Patient guidance
- Breast Augmentation Consultation — Patient guidance
- Questions to Ask Your Plastic Surgeon — Question checklist
- Breast Augmentation Cost — Cost guidance
- Types of Breast Implants — Device overview
- Breast Augmentation Risks and Safety — Safety guidance
- Breast Augmentation Preparation — Pre-operative guidance
- Breast Augmentation Procedure Steps — Procedure steps
- Breast Augmentation Recovery — Recovery guidance

- Breast Augmentation Results — Results guidance
- Choosing a Plastic Surgeon — Credential guidance
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- BAAPS Consumer Safety Guidelines — Safety guidance

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https://www.ncbi.nlm.nih.gov/books/ · captured 2026-10-20

- Continuing Education Activity — StatPearls review section · 2023-07-18
- Introduction — StatPearls review section · 2023-07-18
- Anatomy and Physiology — StatPearls review section · 2023-07-18
- Indications — StatPearls review section · 2023-07-18
- Contraindications — StatPearls review section · 2023-07-18
- Equipment — StatPearls review section · 2023-07-18
- Personnel — StatPearls review section · 2023-07-18
- Preparation — StatPearls review section · 2023-07-18
- Technique or Treatment — StatPearls review section · 2023-07-18
- Complications — StatPearls review section · 2023-07-18
- Clinical Significance — StatPearls review section · 2023-07-18
- Enhancing Healthcare Team Outcomes — StatPearls review section · 2023-07-18
- References and Further Reading — StatPearls reference list · 2023-07-18

Wikipedia

https://en.wikipedia.org/wiki/Breast_augmentation · captured 2026-10-20

- Saline breast prosthesis — Article section
- Silicone-gel breast prosthesis — Article section
- Alternative-composition breast prosthesis — Article section
- Structured breast prosthesis — Article section
- Types of surgical incision — Article section
- Implant-pocket placement — Article section
- Post-surgical recovery — Article section
- Medical complications — Article section
- Rupture of the prosthetic breast — Article section
- Capsular contracture — Article section
- Systemic sickness — Article section
- Fat-graft breast augmentation — Article section

- Fat grafting techniques — Article section
- External tissue expansion — Article section
- Post-mastectomy procedures — Article section
- Tissue engineering: the breast mound and explantation deformity — Article section
- Fat-graft augmentation outcomes and limitations — Article section
- Breastfeeding and the prosthetic breast — Article section
- Breast-filler toxicity and platinum toxicity — Article section
- Psychology of breast-augmentation patients — Article section
- Mental health and mortality findings — Article section

MedlinePlus Medical Encyclopedia (U.S. National Library of Medicine)

https://medlineplus.gov/ · captured 2026-10-20

- Breast augmentation surgery — Medical Encyclopedia article · 2025-02-17
- Cosmetic breast surgery - discharge — Patient instructions · 2025-02-17
- Surgical wound care - open — Patient instructions · 2026-01-01
- Breast lift — Medical Encyclopedia article
- Breast reduction — Medical Encyclopedia article
- Breast augmentation - series — Medical Encyclopedia image series
- Breast lift (mastopexy) - series — Medical Encyclopedia image series
- Breast reduction (mammoplasty) - series — Medical Encyclopedia image series
- Breast lift (anatomy video) — Anatomy video
- Plastic and Cosmetic Surgery — Health topic page · 2024-12-17

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- Wound drainage after plastic and reconstructive surgery of the breast — Cochrane Review · 21 October 2015
- Different types of implants for reconstructive breast surgery — Cochrane Review · 16 May 2016
- Implants versus autologous tissue flaps for breast reconstruction following mastectomy — Cochrane Review · 31 October 2024
- Breast implant insertion funnels for preventing capsular contracture — Cochrane Protocol · 30 March 2026
- Mesh for immediate implant-based prepectoral breast reconstruction following mastectomy — Cochrane Protocol · 17 June 2025
- Acellular dermal matrices for breast reconstruction surgery — Cochrane Protocol · 2015
- Autologous fat grafting for breast reconstruction — Cochrane Protocol · 29 June 2021

How to use a bibliography

Listing a source here means it forms part of the evidence base for a topic — not that its every claim is endorsed or current. Check the source's date, its origin and whether it describes your jurisdiction before relying on it.

Your journey starts with information

Not sure how to read a source?

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss

them with a qualified, registered healthcare professional who can assess you in person.

ASPS Breast Augmentation Information | Breast Implants, Costs & Safety | Breast Enlargement Guide

Page: /research/asps-breast-enlargement/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Professional society library

ASPS Breast Augmentation Information

Breast augmentation information published by the American Society of Plastic Surgeons — what the procedure is, candidates, consultation, costs, implant types, safety, preparation, recovery, results and choosing a surgeon, extracted and linked to the original plasticsurgery.org pages.

This library collects the American Society of Plastic Surgeons' public patient information on breast enlargement: the overview page, candidacy, consultation, questions to ask, cost, implant types, risks and safety, preparation, procedure steps, recovery, results, choosing a surgeon and fat transfer. Every entry links to the original page on plasticsurgery.org , so the wording and any subsequent updates can be checked at source.

What this library does and does not mean

ASPS is a professional membership organization for board-certified plastic surgeons, not a government regulator and not a clinic. Its pages describe breast augmentation as performed by its member surgeons and the standards it expects of them. Nothing here is a recommendation to have a procedure, and ASPS does not endorse any individual surgeon, clinic or product.

Summaries and key points are condensed from ASPS pages for readability. They are not a substitute for the original documents, which may have been updated since this library was compiled on 2026-10-20. Always read the current version on plasticsurgery.org and discuss it with a qualified clinician.

Coverage

What the ASPS information covers

The library is grouped into six areas, from the overview of the operation to devices, safety, aftercare, fat transfer and the surgeon credentials to check.

Decisions and consent

What augmentation can and cannot do, who makes a candidate, what happens at consultation and the questions the ASPS checklist tells patients to ask.

Implants and techniques

Saline, silicone and structured fills, form-stable, round, smooth and textured surfaces, incision and placement options, and the five procedure steps.

Risks and safety

Capsular contracture, rupture, BIA-ALCL, BIA-SCC, breast implant illness, the fact that implants are not lifetime devices and the FDA screening intervals.

Preparation and recovery

Medication and substance changes before surgery, accredited facilities, support at home, discharge timing, pain

timeline and how results settle.

Fat transfer

Using your own fat harvested by liposuction for a smaller, natural-feeling increase, and the ASPS-cited research behind the technique.

Cost and credentials

Average surgeon fees for implant and fat-grafting augmentation, what those figures exclude, insurance position, board certification and facility standards.

All ASPS pages in this library

Grouped by topic. Each entry gives a short summary, the main extracted points and a direct link to the full ASPS page.

Breast augmentation

The ASPS overview of breast augmentation: what the operation is, what it can and cannot do, who makes a suitable candidate, what happens at consultation and the questions patients are advised to ask.

Breast Augmentation (Augmentation Mammaplasty)

Procedure overview

The ASPS overview of breast augmentation: a procedure that uses breast implants or fat transfer to increase breast size. It explains that augmentation can restore volume lost after weight reduction or pregnancy, create a rounder shape or correct natural asymmetry, lists what the surgery can and cannot achieve, and points patients to implant safety, fat transfer and breast lift information.

- Breast augmentation (also called augmentation mammaplasty) uses breast implants or fat transfer to increase the size of the breasts.
- It can restore volume lost after weight reduction or pregnancy, achieve a rounder shape, or improve natural size asymmetry.
- When fat from another part of the body is used, the procedure is called fat transfer breast augmentation.
- Surgery can increase fullness and projection, improve the balance of breast and hip contours, and enhance self-image and confidence.
- Augmentation does not correct severely drooping breasts; a breast lift may be required alongside it for sagging breasts to look fuller and lifted.
- A lift can often be done at the same time as augmentation, or may require a separate operation.
- Implants are also used for breast reconstruction after mastectomy or injury.

Read the original on plasticsurgery.org →

Breast Augmentation Candidates

Patient guidance

ASPS guidance on who may be a candidate for breast augmentation. The page stresses that the procedure is deeply personal and should be chosen for oneself, lists the medical and developmental factors surgeons look for, and describes the common reasons people seek augmentation.

- The decision is deeply personal and should be made for yourself rather than to satisfy someone else.
- Candidates should be physically healthy and not pregnant or breastfeeding.
- Breasts should be fully developed and expectations should be realistic.
- Common motivations include dissatisfaction with small size, loss of shape or volume after pregnancy or weight loss, and changes with aging.

- Other reasons include an empty upper breast, asymmetry, or one or both breasts failing to develop normally.
- Reviewing before-and-after photos and recovery information beforehand helps set reasonable expectations.

Read the original on plasticsurgery.org →

Breast Augmentation Consultation

Patient guidance

What to expect at a breast augmentation consultation: the topics patients should be ready to discuss, the medical history the surgeon collects, and the physical assessment and measurements used to make a recommendation.

- Be ready to discuss your motivation, expectations and the outcome you want.
- Disclose medical conditions, allergies, previous treatments, medications, supplements and any substance use.
- Share family history of breast cancer and the results of previous mammograms or biopsies.
- The surgeon evaluates general health and risk factors, then takes detailed breast measurements and photographs.
- The examination covers size, shape, skin quality and nipple-areola placement before treatment is recommended.
- The surgeon discusses the expected outcome, risks and complications, and offers a written question checklist.

Read the original on plasticsurgery.org →

Questions to Ask Your Plastic Surgeon

Question checklist

The ASPS checklist of questions patients should take to a breast augmentation consultation, covering surgeon credentials, facility accreditation, the surgical plan, recovery support, risk handling and long-term considerations.

- Verify board certification by the American Board of Plastic Surgery and plastic-surgery-specific training.
- Ask about years of surgical training and where the surgeon has hospital privileges.
- Confirm the surgical facility is accredited, state-licensed or Medicare-certified.
- Discuss candidacy, the recommended implant shape, size and texture, and incision and placement choices.
- Ask how long recovery takes, what help you will need, the risks, and how complications would be handled.
- Ask about possible future operations, effects on breastfeeding, and how results change over time and after pregnancy.

Read the original on plasticsurgery.org →

Breast implants

ASPS information on the implants themselves: fill types (saline, silicone, structured), shapes and surfaces (form-stable, round, smooth, textured), FDA age approvals and surface-related safety concerns.

Types of Breast Implants

Device overview

ASPS guidance on the main implant options: fill type (saline, silicone gel, structured saline) and shape or surface (form-stable "gummy bear", round, smooth, textured), with the FDA age approvals and the main trade-offs of each choice.

- Saline implants are filled with sterile salt water; if they leak the implant collapses and the body safely absorbs the fluid.
- Silicone gel implants are designed to mimic the feel of natural tissue; a leak may stay contained and the implant does not collapse.
- Structured saline implants contain an inner matrix intended to give a more natural feel.

- Form-stable "gummy bear" implants keep their shape even if the shell breaks; they use a firmer gel, need a longer incision and shaped versions can rotate.
- Round implants look fuller and do not rotate out of place; smooth implants feel softest but may ripple, while textured implants grip tissue.
- Textured surfaces are the surface most associated with breast implant-associated anaplastic large cell lymphoma (BIA-ALCL).

Read the original on plasticsurgery.org →

Risks and safety

ASPS information on the risks of augmentation and implant-specific safety concerns, including capsular contracture, rupture, BIA-ALCL, BIA-SCC, breast implant illness and FDA screening intervals.

Breast Augmentation Risks and Safety

Safety guidance

The full list of risks ASPS publishes for breast augmentation — from anesthesia complications and capsular contracture to implant-associated cancers and breast implant illness — together with the reminder that implants are not lifetime devices and that FDA-recommended screening applies to silicone implants.

- Common risks include anesthesia complications, bleeding or hematoma, infection, capsular contracture, implant rupture, changes in sensation, poor scarring and possible revision surgery.
- Rare cancers of the implant capsule (BIA-ALCL and BIA-SCC) and systemic symptoms described as breast implant illness are acknowledged.
- Most people with implants never experience serious complications, but FDA-approved devices still carry documented risks.
- Implants are not guaranteed for life; future replacement surgery may be needed.
- Ongoing breast-health exams and implant-condition checks are needed, and pregnancy, weight loss and menopause change appearance over time.
- Routine screening matters because some ruptures produce no symptoms.

Read the original on plasticsurgery.org →

Preparation, procedure and recovery

Practical steps around the operation itself: pre-operative preparation, the five surgical steps (anaesthesia, incision, placement, closure, result), immediate aftercare and the recovery timeline.

Breast Augmentation Preparation

Pre-operative guidance

The medical and lifestyle steps ASPS advises before breast augmentation: tests and medication changes, stopping substances that increase bleeding, choosing an accredited surgical facility and arranging post-operative support.

- You may need blood tests and adjustments to your medications, including stopping certain drugs.
- Stop smoking and avoid aspirin and anti-inflammatory drugs that can increase bleeding.
- Recreational drugs such as cocaine must be stopped before surgery.
- Breast augmentation should be performed in an accredited outpatient or ambulatory surgical center or a hospital.
- If going home the same day, arrange a driver and someone to stay with you for at least the first night.

Read the original on plasticsurgery.org →

Breast Augmentation Procedure Steps

Procedure steps

The five steps of breast augmentation as set out by ASPS — anaesthesia, incision, implant placement, closing the incisions and the immediate result — with the incision and placement options and a caution about the transumbilical approach.

- Step 1: anesthesia with IV sedation or general anesthesia, selected for the patient.
- Step 2: incisions placed in discreet locations — around the areola (peri-areolar), in the breast fold (inframammary) or in the armpit (axillary).
- Step 3: the implant is placed either under the pectoral muscle (submuscular) or behind the breast tissue over the muscle (submammary).
- Step 4: incisions are closed with layered sutures plus skin sutures, adhesive or tape; scar quality depends on genetics, nicotine use and infection.
- Step 5: results are visible immediately after surgery.
- The belly-button (transumbilical) incision approach is associated with a higher complication rate.

Read the original on plasticsurgery.org →

Breast Augmentation Recovery

Recovery guidance

What to expect straight after breast augmentation: dressings and support garments, monitoring and discharge timing, the instructions and medications you go home with, and the pain and activity timeline during healing.

- Breasts are wrapped in gauze with an elastic bandage or support bra to limit swelling.
- Patients are monitored in recovery and typically discharged about an hour after surgery once stable.
- You receive written post-operative instructions, a follow-up appointment and possibly pain medication and an antibiotic.
- The support garment or bandeau should be worn around the clock as directed; incision-care instructions vary by surgeon.
- Take it easy for the first several days because of bleeding risk around the implant pockets, and resume exercise only when cleared.
- Acute pain usually subsides after one to five days, while soreness and swelling may persist for a few weeks.

Read the original on plasticsurgery.org →

Breast Augmentation Results

Results guidance

How breast augmentation results settle over time: shape finalising over the first weeks as swelling resolves, incision marks fading slowly, the follow-up cadence, and the fact that implants are not lifetime devices and results change with aging.

- Breasts are larger right away, but the final shape takes a few weeks as swelling subsides and the skin stretches.
- Some patients, such as those with asymmetry or very small starting breasts, may need a bandeau to help shape the result.
- Incision lines can take months — even a couple of years — to fade.
- Optimal results depend on following post-operative instructions and attending follow-up visits.
- Implants are not lifetime devices; annual exams evaluate breast health and implant integrity.
- Aging, weight fluctuation, hormones and gravity change the result; some patients later choose a lift or implant exchange.

Read the original on plasticsurgery.org →

Fat transfer breast augmentation

ASPS information on enlarging the breasts with your own fat: liposuction harvesting, reinjection, who it suits and the research it references.

Fat Transfer Breast Augmentation

Procedure overview

The ASPS page on fat transfer breast augmentation: liposuction is used to harvest fat from elsewhere in the body and reinject it into the breasts, offered as an option for people wanting a relatively small, natural-looking increase in size.

- Fat is taken by liposuction from other parts of the body and injected into the breasts to enhance them.
- It is an option for people looking for a relatively small increase in breast size.
- It is described as a choice for those who would prefer natural results.
- ASPS cites research on pre-expansion for mega-volume transfer, fat transfer as an alternative to implants, and techniques that substantially increase breast size using the patient's own fat.
- When fat transfer is used, the procedure is referred to as fat transfer breast augmentation rather than augmentation mammaplasty with implants.

Read the original on plasticsurgery.org →

Cost and choosing a surgeon

What ASPS publishes on augmentation fees, what is and is not included, insurance coverage, and the credentials an ASPS member plastic surgeon must hold.

Breast Augmentation Cost

Cost guidance

ASPS guidance on the cost of breast augmentation, including its published average surgeon fees for implant and fat-grafting augmentation, the expenses those figures exclude, and the insurance position for cosmetic surgery.

- Published average costs reflect surgeon fees only, not the total price of the procedure.
- Anesthesia, operating-room or facility fees, medical tests, garments, prescriptions and related expenses are excluded.
- The surgeon fee varies with experience, procedure type and geographic location.
- Many surgeons offer patient financing plans.
- Most insurance plans do not cover cosmetic augmentation, related complications or revision surgery.
- Some carriers exclude breast-disease coverage for implant patients.

Read the original on plasticsurgery.org →

Choosing a Plastic Surgeon

Credential guidance

The credentials ASPS says patients should look for: an ASPS member plastic surgeon board-certified by the American Board of Plastic Surgery (or the Royal College of Physicians and Surgeons of Canada), with the required training, exams, continuing education and accredited facility standards.

- Choose an ASPS member plastic surgeon board-certified by the American Board of Plastic Surgery or the Royal College of Physicians and Surgeons of Canada.
- Certification requires at least six years of surgical training after medical school, including a minimum of three years of plastic surgery residency.
- Surgeons must pass comprehensive oral and written exams and graduate from an accredited medical school.
- Continuing medical education including patient safety must be completed every year.
- Surgery must be performed in accredited, state-licensed or Medicare-certified facilities.

- The ABPS is recognized by the ABMS, and no ABMS-recognized certifying board has "cosmetic surgery" in its name.

Read the original on plasticsurgery.org →

Extracted data

Key facts from the ASPS pages

The statements below summarise what ASPS says about breast enlargement and related decisions. The right-hand column links to the source page.

Topic What ASPS says Source Average cost with implants ASPS reports an average surgeon fee of $4,875 for breast augmentation with implants. Breast Augmentation Cost Average cost with fat grafting ASPS reports an average surgeon fee of $5,719 for augmentation by fat grafting. Breast Augmentation Cost What the average covers Quoted averages are surgeon fees only; anesthesia, facility fees, tests, garments and prescriptions are excluded. Breast Augmentation Cost Insurance cover Most insurance plans do not cover cosmetic augmentation, its complications or revision surgery. Breast Augmentation Cost Saline age approval The FDA approves saline implants for augmentation in women aged 18 or older. Types of Breast Implants Silicone age approval The FDA approves silicone implants for augmentation in women aged 22 or older. Types of Breast Implants Textured surface and BIA-ALCL Breast implant-associated anaplastic large cell lymphoma occurs most frequently with textured-surface implants. Breast Augmentation Risks and Safety First silicone screening The FDA recommends MRI or ultrasound screening 5-6 years after silicone implant placement. Breast Augmentation Risks and Safety Ongoing silicone screening After the first screen, the FDA recommends screening every 2-3 years. Breast Augmentation Risks and Safety Silent rupture Some women with a ruptured implant have no symptoms, which is why routine screening is advised. Breast Augmentation Risks and Safety Implant longevity Implants are not guaranteed for life and future replacement surgery may be needed. Breast Augmentation Risks and Safety Facility standard Augmentation should be performed in an accredited outpatient or ambulatory surgical center or a hospital. Breast Augmentation Preparation Substances and bleeding Smoking, aspirin, anti-inflammatory drugs and recreational drugs such as cocaine should be stopped before surgery because they increase bleeding. Breast Augmentation Preparation Anesthesia The procedure is performed under IV sedation or general anesthesia. Breast Augmentation Procedure Steps Incision options Incisions may be peri-areolar, in the inframammary fold or in the armpit (axillary). Breast Augmentation Procedure Steps Placement options Implants are placed either under the pectoral muscle or behind the breast tissue over the muscle. Breast Augmentation Procedure Steps Transumbilical approach The belly-button incision approach is associated with a higher complication rate. Breast Augmentation Procedure Steps Discharge timing Patients typically go home about an hour after surgery once they are stable. Breast Augmentation Recovery Acute pain Acute pain usually subsides after 1-5 days. Breast Augmentation Recovery Soreness and swelling Soreness and swelling may persist for a few weeks after surgery. Breast Augmentation Recovery Final result timeline Final results may take a few weeks to appear as swelling subsides and the skin stretches. Breast Augmentation Results Scar maturation Incision lines can take several months, up to a couple of years, to fade. Breast Augmentation Results Follow-up Annual examinations are suggested to assess breast health and implant integrity. Breast Augmentation Results Surgeon training ASPS member surgeons complete at least six years of surgical training after medical school, including at least three years of plastic surgery residency. Choosing a Plastic Surgeon Board certification Patients are advised to verify certification by the American Board of Plastic Surgery, which is recognized by the ABMS. Choosing a Plastic Surgeon Facility credentials ASPS member surgeons operate in accredited, state-licensed or Medicare-certified surgical facilities. Choosing a Plastic Surgeon Fat transfer method Fat transfer augmentation uses liposuction to take fat from elsewhere in the body and inject it into the breasts. Fat Transfer Breast Augmentation Fat transfer suitability Fat transfer is presented as an option for a relatively small increase in size for those wanting natural results. Fat Transfer Breast Augmentation Statements are paraphrased from ASPS patient information. Verify against the linked plasticsurgery.org page before relying on any point, and note that fee figures are national averages reported by ASPS.

Jurisdiction and copyright

ASPS pages describe practice, board certification and fees in the United States. Board-certification bodies, FDA device approvals, facility accreditation and insurance rules are US arrangements, and standards differ between countries.

This library stores only summaries, extracted facts and links to publicly available ASPS pages. Website content of the American Society of Plastic Surgeons is their property and is subject to their terms and conditions .

For regulator guidance, see the FDA information library . For UK patient information, see the NHS information library . For Australian Government health advice, see the healthdirect information library . For academic patient education, see the Cleveland Clinic information library . For published research, see the PMC research library .

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Your journey starts with information

Want to discuss what the ASPS guidance means for you?

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

BAAPS Breast Enlargement Information | Augmentation Guide, Implant Safety & Consumer Advice | Breast Enlargement Guide

Page: /research/baaps-breast-enlargement/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Association library

BAAPS Breast Enlargement Information

Breast enlargement information published by the British Association of Aesthetic Plastic Surgeons — the joint ABS, BAAPS and BAPRAS patient booklet, breast implant safety and consumer safety guidance, extracted and linked to the original baaps.org.uk pages.

This library collects the BAAPS's public patient information on breast enlargement and the decisions around it — including the joint ABS, BAAPS and BAPRAS booklet Your Guide to Breast Augmentation . Every entry links to the original page or PDF on baaps.org.uk , so the wording, dates and any subsequent updates can be checked at source.

What this library does and does not mean

BAAPS is a professional association of aesthetic plastic surgeons; it publishes patient education and safety guidance but does not endorse any particular clinic, surgeon outside its membership, or treatment package. Nothing here is a recommendation to have a procedure, and individual suitability always requires assessment by a qualified clinician.

Summaries and key points are condensed from BAAPS pages and the joint patient booklet for readability. They are not a substitute for the original documents, which may have been updated since this library was compiled on 2026-10-20. Always read the current version on baaps.org.uk and discuss it with a qualified clinician.

Coverage

What the BAAPS information covers

The library is grouped into four areas, from the procedure itself to implant safety and consumer guidance.

The procedure

What breast augmentation involves, consultation and consent, implant placement, operation time, recovery and expected outcomes.

The patient booklet

The joint ABS, BAAPS and BAPRAS guide: decisions, costs, techniques, implant type, size, shape and texture, complications with frequencies, long-term consequences and alternatives.

Implant safety

Breast Implant Illness (BII), BIA-ALCL, BIA-SCC, PIP implants and the association's published safety statements.

Consumer guidance

Fourteen safety points covering your own decision, research, questions, credentials, cooling-off periods, timing and taking your time.

Complications

Bleeding, infection, asymmetry, capsular contracture, sensation changes, palpability, rupture and revision rates with typical frequencies from the booklet.

Alternatives

Fat transfer (lipofilling) and breast uplift (mastopexy) as alternatives or adjuncts to implant augmentation, and the UK implant registry.

All BAAPS sources in this library

Grouped by topic. Each entry gives a short summary, the main extracted points and a direct link to the full BAAPS page or PDF.

Breast augmentation

BAAPS patient information on breast enlargement itself: what the operation involves, consultation and consent, techniques, implant choices, recovery and expected outcomes.

Breast Augmentation

Procedure page · Updated 2021

The BAAPS procedure page for breast augmentation (bilateral breast augmentation, BBA) — an operation to enlarge the breasts, usually by inserting an implant beneath the breast tissue. The page describes the procedure as potentially life-enhancing but stresses that it is a big decision, major surgery with potential risks, and that the hoped-for result cannot be guaranteed. It links to the full joint patient booklet and lists the common names patients use for the procedure.

- Breast augmentation (BBA) is an operation to enlarge the breasts, usually by inserting an implant beneath the breast tissue.
- It can be a life-enhancing procedure, but having a breast enlargement is a big decision.
- It is major surgery with potential risks, and the result you may hope for cannot be guaranteed.
- The joint ABS, BAAPS and BAPRAS booklet explains the procedure and the short- and long-term risks; any further questions after consultation should be asked of your surgeon.
- The procedure is also commonly known as a boob job, enlargement, implant or breast size.

Read the original on baaps.org.uk →

Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS)

The joint patient booklet from the Association of Breast Surgery, BAAPS and BAPRAS (Edition 2, 2021): a section-by-section guide covering decisions, surgery, implants, complications, long-term consequences, alternatives and the national implant registry.

Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021)

Patient information booklet · Updated 2021-04

The joint patient booklet from the Association of Breast Surgery, BAAPS and BAPRAS. It covers who the guide is for and what breast augmentation involves; how to seek a consultation, check a surgeon's qualifications, understand costs and the cooling-off period; the surgery itself including implant placement (sub-glandular, sub-muscular, dual plane), operation time and recovery; implant type, size, shape and texture; a full list of complications with typical frequencies; long-term consequences including mammography, BIA-ALCL and breast implant illness; alternatives such as lipofilling and mastopexy; and the UK Breast and Cosmetic Implant Registry. Editors: Lee Martin and Emma de Sousa; reviewed by PRASEAG.

- Breast augmentation is usually done by inserting an implant beneath the breast to make it look larger; it can enlarge always-small breasts or fill out breasts that have emptied, for example after pregnancy.

- It will reshape breasts to some extent and can also be used to correct unequal breast size.
- Fat transfer (lipomodelling, lipofilling or fat grafting) is an alternative in a few people but is not suitable for all and often requires more than one episode of surgery.
- Patients tend to be pleased with results, but you need realistic expectations and must understand what can and cannot be achieved, the limitations of surgery and the long-term consequences of implants.
- Your initial consultation should be with the surgeon who will do the operation; your surgeon will normally seek permission to write to your GP; a consultation fee is normal.
- At consultation you will be examined, measurements taken, and implant shapes, sizes and pocket positions discussed; some surgeons use implant sizers in a bra or 3D photography, which is only a guide — the same implant can look very different in different people.
- You should be given a cooling-off period of at least two weeks before surgery and offered a second consultation; you should not feel rushed.
- Breast augmentation is a lifetime commitment, not a one-stop permanent solution: you will need revision surgery as the years go by and must be prepared personally and financially.
- Nobody needs an urgent breast augmentation — if you are not offered a cooling-off period or are put under pressure, walk away.
- Surgeons should be listed on the GMC specialist register (gmc-uk.org); membership of ABS, BAAPS and/or BAPRAS indicates a standard set of recognised qualifications.
- Internet search listings may be promotional; a prominent listing measures marketing ability, not service quality.
- Cosmetic breast augmentation is not available on the NHS except in exceptional circumstances such as very uneven breasts, no breasts, or as part of reconstruction; after surgery abroad the NHS will only deal with emergency complications on return.
- Breast enlargement involves placing an implant under the breast tissue; implants are usually inserted through an incision under the breast at the crease, but can also go in via the armpit or around the nipple.
- Implants can sit behind the breast on top of the chest wall muscle (sub-glandular) or behind the breast and muscle (sub-muscular); your surgeon will advise which is appropriate.
- Sub-glandular placement is considered the simplest enlargement procedure and less likely to cause significant discomfort; it is also effective with some drooping.
- Sub-muscular placement provides more padding for the implant, helpful for slender patients and those with little breast tissue, reducing the chances of feeling or seeing the implant.
- Dual plane augmentation combines the two routes, placing implants partly behind the breast and partly behind the muscle; implants behind the muscle may move with muscle contraction (animation), which is normal.
- Breast enlargement surgery takes about one-and-a-half hours, usually under general anaesthetic; you may go home the same day but many patients spend one night in hospital.
- You will be mobile from day one and should be back to full exercise within six weeks; one to two weeks off work is recommended.
- At first the breasts may look high and the skin tight; this tends to settle over the first six weeks, with the long-term result emerging over 3-4 months as a more natural shape appears.
- Scars tend to be red in the first six weeks, change to purple over the next three months, then fade to white; abnormal scarring can occur.
- All implant shells are made of silicone; some have an additional polyurethane coating; shells are filled with silicone gel or saline, and implants have been used since the 1960s with evidence suggesting they are safe.
- Most patients are offered silicone gel-filled implants — they feel most natural, have a range of shapes and are durable; saline implants tend to feel less natural, ripples may be more visible and they carry a risk of deflation; all other filler materials have been withdrawn for use in the UK.
- Implants are supplied by volume and it is not possible to guarantee a cup size; the larger the implant and the slimmer the patient, the less natural the result, and an out-of-proportion implant is more likely to cause drooping, tissue stretching and palpability.
- Implants are round or teardrop-shaped (anatomical); round implants give bigger volume at the top and are the

same width as tall, while teardrop implants allow width and height to be chosen separately; both come in varying degrees of projection.

- Texturing reduces capsular contracture (particularly with implants placed in front of the muscle) and reduces movement or rotation; anatomical implants tend to be textured because rotation would alter the shape, while rotation of a round implant is not noticeable.
- Textured surfaces are related to BIA-ALCL, an uncommon condition; smooth-surface implants currently appear to have no link to BIA-ALCL; smooth implants are almost always round and can take several months to settle, and may slip sideways (lateral displacement) or drop below the crease (bottoming out).
- Small quantities of silicone from implants can be taken up by the body and found in breast tissue and sometimes armpit lymph glands; silicone granuloma is inflammation and extra tissue around silicone deposits.
- Over time the implant shell may fail and gel may leak; many ruptures are discovered by accident, and a ruptured implant should be removed — replacement at the same or a later operation is a choice.
- There is no association between breast augmentation and breast cancer.
- Serious complications are uncommon, occurring in less than 1 in 100 operations.
- Bleeding (haematoma) occurs in less than 1 in 100, usually immediately but occasionally up to two weeks later; the breast becomes very swollen and tight and you return to theatre to have the blood removed — haematoma can increase the risk of later capsular contracture.
- Infection occurs in less than 1 in 1000 cases, usually apparent over the first two or three weeks; most patients with an established infection around the implant need the implant removed, and a new implant cannot be inserted immediately — three to six months should wait for the infection to resolve.
- Perfect symmetry is not an achievable goal; asymmetry can be of breast or nipple, size, shape and position, may appear more pronounced after surgery, and implants can settle differently in their pockets.
- Some patients have a naturally wide cleavage that implants will not improve; with sub-muscular placement the cleavage can appear wider, and breasts and implants may fall to the sides when lying down, which is normal.
- Stretch marks may develop, especially with larger implants, and veins may become more noticeable.
- Breasts will sag with time because they are larger and heavier; the larger the implant, the more the breast will droop, and breasts may bottom out.
- Implants do not interfere with the ability to breastfeed, though some evidence suggests milk production may be reduced in some women; there is no evidence of increased illness in children of women with silicone gel implants.
- Every patient forms a scar capsule around the implant; capsular contracture — when this scar contracts and makes the implant feel firmer — is up to 1 in 10 of all augmentations, but most of those patients will not need revision surgery.
- About 1% (1 in 100) a year need re-operation for any reason, so after 10 years about 10% will have had more surgery; capsular contracture is the most common reason and is likely to recur in 1 in 2 cases after secondary surgery.
- Around 1 in 5 patients will have reduced nipple sensation; recovery can take up to 12 months, but 1 in 10 will have some permanent numbness.
- It is common to be able to feel the implant, especially in slim patients or those with little breast tissue; this is an inevitable consequence of the operation and will not improve with time; lipomodelling can improve the situation in some patients.
- The American Core study (FDA Update 2011) suggested 1 in 10 implants had ruptured at 10 years, and half of all implants may rupture by 15 years; many ruptures are silent, contained within the capsule, and do not appear harmful.
- There is no universally agreed replacement schedule for implants and it is unusual to need exchange before ten years; if you have noticed no change you do not need regular follow-up or scans — but you should be prepared personally and financially for possible future surgery.
- Other reasons for re-operation include wanting larger implants, shape change with time or after pregnancy and weight fluctuation, rotation of teardrop implants, and intermittent swelling around implants.
- Long-term problems will not usually be covered by the package bought at the time of augmentation, and the

NHS should not be expected to provide future private treatment.

- BIA-ALCL is recognised by the WHO as a condition associated with breast implants; it is not a breast cancer but a cancer of the scar tissue capsule, occurring between 2 and 28 years after insertion (average 8 years), usually presenting as swelling around the implant (seroma), and usually treated by removing the implant and capsule.
- Most BIA-ALCL cases worldwide have occurred with textured implants, with higher numbers where the texture is coarser; breast implants continue to have safety approval from the MHRA and FDA and continue to be used in reconstruction worldwide.
- Breast Implant Illness (BII) is a term used by some patients for symptoms including tiredness, brain fog, joint aches, immune-related symptoms, sleep disturbance, depression, hormonal issues, headaches, hair loss, chills, rash and neurological issues; it is not a medical diagnosis and there is no proven association with implants.
- Some patients report symptoms improve if implants are removed, but this is not true for all; research continues.
- Implants do not make breast self-examination more difficult, but they do interfere with mammography because X-rays cannot pass through the implant; tell the mammography service you have implants — special views or a different centre may be used, and some patients opt for private MRI screening.
- It is safe to sunbathe and fly with implants; implant massage is not needed and is not recommended with modern textured implants; routine implant replacement after a specified time is not recommended.
- The only alternative surgical technique to enlarge the breast is lipofilling: fat removed by liposuction from hips, thighs or tummy and injected into the breast; only a relatively small amount can be injected so multiple operations are needed for a worthwhile effect, and some fat is absorbed in the initial weeks — fat that lasts brings permanent enlargement.
- Your own fat is the only substance that can be safely injected into the breast; other materials have been tried and withdrawn — do not allow anyone to inject anything other than your own fat.
- Breast uplift (mastopexy) is a different operation that lifts the nipple and breast tissue and tightens the skin without an implant; scarring is more obvious, and uplifted breasts tend to droop again with time and gravity.
- Mild droopiness can be improved by augmentation alone, but where volume loss and droopiness both bother you, augmentation alone will not solve drooping — your surgeon should discuss augmentation with mastopexy, as one operation or in two stages.
- In the UK the national Breast and Cosmetic Implant Registry records all implants placed or removed, allowing patients to be traced in the event of a future recall and identification of trends; entry requires consent and is strongly recommended.
- Avoid any deal with a non-refundable deposit or financial inducement to proceed — even refunding a consultation fee if you proceed with surgery is considered an inducement and against GMC guidance.
- A package price covering the entire process and the cost of treating complications in the initial weeks is appropriate; the cheapest deal may not be the best.
- Before your consultation it should be possible to obtain an approximate cost from your private hospital or the surgeon's secretary; after consultation(s) you should be given a written quote.

Read the original on baaps.org.uk →

Breast implant safety

BAAPS information on breast implant associated conditions — Breast Implant Illness (BII), BIA-ALCL, BIA-SCC, PIP implants — and the association's published safety statements.

Breast Implant Safety

Safety information page

BAAPS information and guidance on breast implant associated conditions, with links to patient documents on Breast Implant Illness (BII) and BIA-ALCL, the association's updated BII statement of October 2022, joint statements on BIA-SCC and other breast implant associated lymphomas, PRASEAG guidance, the Allergan Natrelle textured implant withdrawal statement, the BAAPS/ABS/BAPRAS joint statement on breast implant safety, and BAAPS advice on PIP implants including signs and symptoms of rupture or inflammation.

- BAAPS publishes patient documents on Breast Implant Illness (BII) and Anaplastic Large Cell Lymphoma (BIA-ALCL), plus an updated BII statement from October 2022.
- In July 2023 BAAPS issued a joint statement on Breast Implant Associated Squamous Cell Carcinoma and other breast implant associated lymphomas.
- PRASEAG issued a statement on breast implants and guidance for patients following MHRA advice on BIA-ALCL, with BAAPS, BAPRAS and the Association of Breast Surgery working together.
- BAAPS issued statements on the withdrawal of Allergan (Natrelle) textured implants and a joint BAAPS/ABS/BAPRAS statement on breast implant safety (27 November 2018).
- On PIP implants the 2013 expert group found no evidence they were responsible for an increased rate of breast cancer; the gel was less cohesive than most contemporary implants and had a greater potential to cause local inflammation on rupture.
- Signs of PIP rupture, leak or inflammation include lumpiness of the breast, lumpiness or swelling of regional lymph nodes, change in shape and size, redness, tenderness, swelling and firmness; symptoms may include pain and hypersensitivity.
- Patients unsure of their implant make should contact their surgeon or provider; patients with signs or symptoms of rupture or irritation should seek advice from their surgeon.
- BAAPS advised removal or replacement of PIP implants as a precaution because of the inferior quality of the gel, regardless of rupture rate.
- Breast implants continue to have safety approval from government organisations such as the UK MHRA and USA FDA and continue to be used in reconstruction worldwide.

Read the original on baaps.org.uk →

Consumer safety guidelines

BAAPS fourteen-point consumer safety guidance for anyone considering aesthetic surgery, from making your own decision to checking credentials and taking your time.

BAAPS Consumer Safety Guidelines

Safety guidance

BAAPS fourteen-point consumer safety guidance for anyone considering aesthetic surgery: make your own decision, do your research, ask the right questions at every stage, know your surgeon and check credentials on the GMC specialist register, beware pressurising selling and non-refundable deposits, expect a two-week cooling-off period, get a second opinion if needed, time the surgery well, think about location and travel, talk to your GP, remember you can change your mind, use common sense about special offers, and take your time.

- Only you can decide whether to go ahead with aesthetic surgery, but you need specialist input to understand what is possible, suitable and the risks and benefits.
- No procedure is 100% risk free and no surgeon can give a 100% guarantee of results; research the limitations and risks before any consultation.
- Know what will happen at every stage — pre-operative assessment, the day of surgery, aftercare, recovery time, follow-up and the possibility of further procedures.
- Your consultation should be with the actual surgeon who will operate; clinic managers and nurses should not be your only point of contact.
- A specialist plastic surgeon should have the FCRS (Plast) qualification and be listed on the Specialist Register of Plastic Surgeons with the GMC.
- Avoid any marketing or selling technique that financially or temporally traps you, including booking fees and non-refundable advances; your decision should be free of incentive.
- You should expect a two-week cooling-off period following your consultation to consider your options.
- Get a second opinion if necessary; a reputable surgeon will never apply a time limit, withhold key information or upsell additional surgery.

- Avoid surgery around major life events such as moving house, changing job, bereavement, relationship breakdown or the arrival of children.
- Give careful consideration to travelling long distances or overseas for surgery; travel insurance often will not cover treatment or extra nights for elective aesthetic procedures.
- Your GP can offer unbiased advice with no vested interest and may discuss functional, psychological and social benefits.
- You can cancel right up until the time you go to sleep for surgery.
- Don't be tempted by special offers or rushed into a quick surgery slot; anything that seems too good to be true probably is.
- Take as much time as you need after an initial consultation; a surgeon might recommend two consultations before booking a procedure.

Read the original on baaps.org.uk →

Extracted data

Key facts from the BAAPS sources

The statements below summarise what BAAPS and the joint booklet say about breast enlargement and related decisions. The right-hand column links to the source page.

Topic What BAAPS says Source What breast augmentation is Breast augmentation (bilateral breast augmentation or BBA) is an operation to enlarge the breasts, usually by inserting an implant beneath the breast tissue; it is also commonly known as a boob job, enlargement, implant or breast size. Breast Augmentation Realistic expectations It can be a life-enhancing procedure, but having a breast enlargement is a big decision: it is major surgery with potential risks, and the result you may hope for cannot be guaranteed. Breast Augmentation Who the booklet is for The joint ABS, BAAPS and BAPRAS booklet is for those looking for more information about breast augmentation, designed to be used alongside discussions with your surgeon, GP, family and friends. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Indications Augmentation will enlarge breasts that have always been small and can fill out breasts that used to be larger, for example after pregnancy; it will reshape breasts to some extent and can correct unequal size. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Consultation Your initial consultation should be with the surgeon who will do the operation; you will be examined and measurements taken, and implant shapes, sizes and pocket positions discussed; sizers or 3D photography are only a guide because the same implant can look very different in different people. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Cooling-off period You should be given a cooling-off period of at least two weeks before surgery and offered a second consultation; nobody needs urgent breast augmentation, and if you are pressured to proceed you should walk away. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Lifetime commitment Breast augmentation is a lifetime commitment and must not be considered a one-stop permanent solution; you will need revision surgery as the years go by and must be prepared personally and financially. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Checking credentials All surgeons should be listed on the GMC specialist register with a licence to practise; surgeons may also be registered with ABS, BAAPS and/or BAPRAS, whose members hold a standard set of recognised qualifications. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Cost and inducements You should avoid non-refundable deposits and financial inducements to proceed — even refunding a consultation fee if you book surgery is against GMC guidance; a written quote should follow consultation, and the cheapest deal may not be the best. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) NHS availability Cosmetic breast augmentation is not available on the NHS except in exceptional circumstances such as very uneven breasts, no breasts, or as part of reconstruction; after surgery abroad the NHS will only deal with emergency complications on return. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Incisions Implants are usually inserted through an incision under the breast at the crease, but can also be put in via an incision in the armpit or around the nipple. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Placement Implants sit either behind the breast on top of the chest wall muscle (sub-glandular) or behind the breast and muscle (sub-muscular); dual plane combines the two; animation of sub-muscular implants with muscle contraction is normal. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Operation and recovery Surgery takes about

one-and-a-half hours under general anaesthetic; many patients spend one night in hospital, are mobile from day one, back to full exercise within six weeks, and should take one to two weeks off work. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Settling and scars Breasts may look high and tight at first, settling over six weeks with the long-term result emerging over 3-4 months; scars are red for six weeks, purple for three months, then fade to white. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Implant materials All implant shells are silicone, filled with silicone gel or saline; most patients are offered silicone gel implants for natural feel and durability; all other filler materials have been withdrawn for use in the UK. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Cup size Implants are supplied by volume and it is not possible to guarantee a cup size; larger implants on a slimmer frame look less natural and are more likely to cause complications such as drooping and tissue stretching. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Shape and texture Implants are round or teardrop-shaped with varying projection; texturing reduces capsular contracture and rotation, anatomical implants tend to be textured, and textured surfaces are related to BIA-ALCL while smooth implants currently appear not to be. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Serious complications Serious complications are uncommon with breast augmentation, occurring in less than 1 in 100 operations. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Bleeding Bleeding (haematoma) risk is less than 1 in 100, usually immediate but occasionally up to two weeks later; return to theatre is needed to remove the blood, and haematoma can increase the risk of later capsular contracture. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Infection Infection occurs in less than 1 in 1000 cases; most established infections around an implant require removal, with three to six months before a new implant can be inserted. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Asymmetry Perfect symmetry is not an achievable goal; asymmetry can involve size, shape, nipple position and implant settling, and may appear more pronounced after surgery. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Capsular contracture Noticeable firmness or capsular contracture occurs in up to 1 in 10 of all augmentations, but most of those patients will not need revision surgery; re-operation for any reason is about 1% a year, around 10% after 10 years, with contracture the most common reason. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Nipple sensation Around 1 in 5 patients will have reduced nipple sensation; recovery can take up to 12 months, but 1 in 10 patients will have some permanent numbness. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Palpability It is common to be able to feel the implant, especially in slim patients or those with little breast tissue; this is an inevitable consequence of the operation and will not improve with time. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Implant rupture The American Core study suggested 1 in 10 implants had ruptured at 10 years and half of all implants may rupture by 15 years; many ruptures are silent and contained within the capsule, but a symptomatic rupture should be investigated and the implant removed. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Replacement schedule There is no universally agreed replacement schedule for breast implants and it is unusual to need exchange before ten years; routine replacement after a specified time is not recommended. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) BIA-ALCL BIA-ALCL is a rare cancer of the implant capsule recognised by the WHO, occurring 2 to 28 years after insertion (average 8 years), usually presenting as swelling around the implant, most often with textured implants — especially coarser textures — and usually treated by removing the implant and capsule. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Breast Implant Illness Breast Implant Illness is a term some patients use for symptoms including tiredness, brain fog, joint aches and others; it is not a medical diagnosis and there is no proven association with implants, though some patients report improvement after removal. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Mammography Implants interfere with mammography because X-rays cannot pass through them; tell the screening service you have implants so special views can be taken, and the more breast volume that is implant the greater the problem; some patients choose private MRI screening. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Breastfeeding and pregnancy Implants do not interfere with the ability to breastfeed, though some evidence suggests milk production may be reduced; pregnancy and breastfeeding may adversely affect the shape of the breast. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Myths It is safe to sunbathe and fly with implants; implant massage is not needed and is not recommended with modern textured implants. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Fat transfer alternative Lipofilling is the only alternative surgical technique to enlarge the breast; only a relatively small amount of fat can be injected so multiple operations are needed, some fat is absorbed early, and fat that lasts brings permanent enlargement — your own fat is the only substance that can be safely injected. Your Guide to Breast Augmentation (ABS,

BAAPS & BAPRAS, Edition 2, 2021) Uplift Mastopexy lifts the nipple and breast tissue and tightens skin without an implant; where both volume loss and droopiness are concerns, augmentation with mastopexy — as one operation or two stages — should be discussed. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Implant registry The UK Breast and Cosmetic Implant Registry records all implants placed or removed so patients can be traced in a future recall; entry requires consent and is strongly recommended for anyone having implant-based surgery. Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021) Implant safety approval Breast implants continue to have safety approval from government organisations such as the UK MHRA and USA FDA and continue to be used in breast reconstruction worldwide. Breast Implant Safety BIA-SCC and other lymphomas In July 2023 BAAPS issued a joint statement on Breast Implant Associated Squamous Cell Carcinoma and other breast implant associated lymphomas. Breast Implant Safety PIP implants The 2013 expert group found no evidence PIP implants increased breast cancer rate, but the gel was less cohesive and had greater potential to cause local inflammation; BAAPS advised removal or replacement as a precaution. Breast Implant Safety PIP warning signs Signs of PIP rupture, leak or inflammation include breast lumpiness, lymph node swelling, change in shape or size, redness, tenderness, swelling and firmness; symptoms may include pain and hypersensitivity. Breast Implant Safety Consumer decision Only you can decide whether to go ahead with aesthetic surgery; no procedure is 100% risk free and no surgeon can give a 100% guarantee of results. BAAPS Consumer Safety Guidelines Cooling-off (consumer guidance) You should expect a two-week cooling-off period following your consultation, and should avoid booking fees, non-refundable advances and any technique that traps you into proceeding. BAAPS Consumer Safety Guidelines Surgeon credentials Your consultation should be with the surgeon who will operate, and a specialist plastic surgeon should have FCRS (Plast) and be on the GMC Specialist Register of Plastic Surgeons. BAAPS Consumer Safety Guidelines Timing and support Avoid surgery around major life events, consider travel and insurance implications carefully, talk to your GP, and take as much time as you need — you can cancel up until the time you go to sleep for surgery. BAAPS Consumer Safety Guidelines Statements are paraphrased from BAAPS patient information and the joint ABS, BAAPS and BAPRAS booklet (Edition 2, 2021). Sources are reviewed and updated periodically; verify against the linked baaps.org.uk page or PDF before relying on any point.

Source and copyright

BAAPS patient information describes practice in the United Kingdom; costs, funding, professional registration and clinical standards differ between countries, and local standards still apply where a procedure is performed. Cosmetic breast augmentation is generally private in the UK; the NHS may fund it only in exceptional circumstances.

The joint booklet Your Guide to Breast Augmentation is published by the Association of Breast Surgery, BAAPS and BAPRAS (Edition 2, BAPRAS London, 2021; editors Lee Martin and Emma de Sousa; reviewed by PRASEAG). This library stores only summaries, extracted facts and links to publicly available BAAPS pages and PDFs; BAAPS content remains the property of BAAPS and the respective publishing bodies.

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Your journey starts with information

Want to discuss what the BAAPS guidance means for you?

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

BAPRAS Breast Augmentation Guide: Implants, Risks & Costs | Breast Enlargement Guide

Page: /research/bapras-breast-augmentation/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Professional association guide

BAPRAS Breast Augmentation Patient Guide

Breast augmentation explained by BAPRAS — the operation, implant choices, the reported risks with their frequencies, the long-term consequences and the decisions you should never be rushed into.

This page works through the patient guide published by the British Association of Plastic, Reconstructive and Aesthetic Surgeons (BAPRAS) — the four-part guide to breast augmentation (bilateral breast augmentation, or BBA): what the operation is, how to choose a surgeon, implant techniques, the complications that can occur and the long-term consequences. Every part is summarised here and linked to the original, so the current wording can always be checked at source.

BAPRAS describes breast enlargement as a potentially life-enhancing procedure — but also as a big decision: major surgery with potential risks, and a result that cannot be guaranteed. The guide was edited by Lee Martin and Emma de Sousa, with contributions from Julie Doughty, Paul Harris, Mark Henley, Nigel Mercer, Charles Nduka, Mary O'Brien, Caroline Payne and Ruth Waters, and was last updated in 2021. A printed copy of the guide is available from the BAPRAS secretariat, and a PDF is published alongside the online version.

This summary is educational. It does not replace the discussion you should have with your own surgeon, and it does not constitute medical advice or a recommendation of any procedure, surgeon or clinic.

The four-part guide

What the BAPRAS guide covers

From the first consultation to the decisions that stay with you for decades.

The decision

Who the guide is for, what breast augmentation can and cannot achieve, and why realistic expectations matter.

Consultation & cooling-off

How to seek a consultation, what happens at it, and why a cooling-off period of at least two weeks is required.

Implant techniques

Incisions, placement behind the breast or muscle, dual plane, and how implant type, size, shape and texture are chosen.

The operation

Anaesthetic, time in hospital, recovery week by week, and how scars typically evolve over the first year.

Complications

Reported frequencies for bleeding, infection, capsular contracture, sensation change and rupture — plus BIA-ALCL and breast implant illness.

Long-term questions

Mammography with implants, alternatives such as lipofilling and uplift surgery, and the national implant registry.

What this page is — and is not

This is a condensed, section-by-section summary of BAPRAS patient information written for people considering breast enlargement in the United Kingdom. Figures such as complication frequencies are those published in the guide; UK practice, funding rules and professional registers differ between countries, so local standards still apply wherever a procedure is performed.

Nothing here is a personal assessment of suitability. That can only be done in a consultation with a qualified, registered surgeon who examines you.

What breast augmentation is

Breast augmentation (bilateral breast augmentation or BBA) is an operation to enlarge the breasts, usually by inserting an implant beneath the breast to make it look larger. It can enlarge breasts that have always been small, and it can also fill out breasts that used to be larger — for example breasts that have emptied out after pregnancy. The procedure reshapes breasts to some extent and can be used to correct breasts that are of unequal size.

An alternative to enlargement with an implant is fat transfer, sometimes called lipomodelling, lipofilling or fat grafting: fat taken from elsewhere on the body is injected into the breast area. Fat transfer may be appropriate for a few people but is not suitable for all, often requires more than one episode of surgery, and can also be combined with an implant augmentation.

Patients who have breast augmentation tend to be pleased with the results, but the guide is explicit that expectations must be realistic. You need to understand what can and cannot be achieved, the limitations of surgery, and the long-term consequences of having breast implants.

Next steps

Getting to a decision

The BAPRAS guide sets out a deliberate, unhurried path from first enquiry to the operating table.

Find a surgeon

Your GP may be able to suggest surgeons locally, or you can approach a private hospital or clinic directly. If you do, make sure your initial consultation is with the surgeon who will actually do the operation. Your surgeon will normally seek permission to write to your GP afterwards.

Attend a full consultation

You will be asked what is bothering you about your breasts and what you expect from surgery, must share your full medical history, and will be examined with measurements taken. Discussion follows about implant shapes, possible sizes and the position of the pockets behind your existing breast tissue. A consultation fee is normal.

See the likely result

Some surgeons use implant sizers in a bra, others 3D photography. This is only a guide — the same implant can look very different in different people. You should be told about the operation, the expected outcome and the possible risks and complications.

Take the cooling-off period

You should be given a cooling-off period of at least two weeks before surgery and offered a second consultation. You should not feel rushed into surgery — if you are, walk away.

Why there is a cooling-off period

Breast augmentation is a lifetime commitment and must not be considered a one-stop permanent solution. You

need to be making the appropriate choice and understand all the implications of the surgery you are considering — including that you will need revision surgery as the years go by, and that you must be prepared personally and financially for this.

Nobody needs an urgent breast augmentation. If you are not offered a cooling-off period, or you are put under pressure to proceed, the guide's advice is simple: walk away.

Checks, costs and funding

Before you commit

Topic What the BAPRAS guide says Checking your surgeon All surgeons should be listed with the General Medical Council (GMC). Check the register at gmc-uk.org using "Check a doctor's registration status". Under "Status" they should be listed as "Registered with a license to practise; this doctor is on the specialist register". Surgeons may also be registered with the Association of Breast Surgery (ABS), BAAPS and/or BAPRAS — members of these organisations hold a standard set of recognised qualifications. It is perfectly reasonable to ask about a surgeon's experience and qualifications. Internet searches Information found through search engines may be promotional in nature; some surgeons and services pay for their listing. A prominent listing is a measure of marketing ability, not of service quality. Cost An approximate cost should be obtainable from your local private hospital or the surgeon's secretary before your consultation, and you should expect to pay a fee for the consultation itself. After your consultation(s), if you plan to go ahead, you should be given a written quote for the planned care. Deposits and inducements Avoid any deal asking for a non-refundable deposit, and do not accept a financial inducement or time-limited deal — even offering to refund the consultation fee if you proceed with surgery is considered an inducement and is against GMC guidance. Package prices It is appropriate to be offered a package covering the whole process and the cost of treating any complications arising in the initial weeks after surgery. You should be told exactly what the package includes. The cheapest deal may not be the best: cosmetic surgery is a significant financial commitment, and you will need to pay for further surgery in the future. NHS funding Cosmetic breast augmentation is not available on the NHS, so you will normally consult and pay privately. Exceptional circumstances can include very uneven breasts (a significant difference in size), having no breasts, or treatment as part of a breast reconstruction care plan — your GP can explain the rules in your area. Surgery abroad If you go abroad for breast augmentation, be aware that on return to the UK only emergency complications will be dealt with by the NHS.
Summarised from the BAPRAS patient guide. Cosmetic-surgery funding, regulation and professional registers differ between countries — verify the current position in your own jurisdiction.

Implant techniques

Breast enlargement involves placing an implant under the breast tissue to increase the size and shape of the breast. Implants are usually inserted through an incision under the breast at the crease, but they can also be placed through an incision in the armpit or around the nipple.

Behind the breast (sub-glandular)

Placing the implant behind the breast, on top of the chest wall muscle, is considered the simplest of the available enlargement procedures and is less likely to cause significant discomfort. This route is also effective for patients with some drooping of the breasts.

Behind the muscle (sub-muscular)

Placing the implant behind the breast muscle gives more padding or coverage for the implant. This can help slender patients and those with very little breast tissue, because it reduces the chances of being able to feel or see the implant under the breast tissue.

Dual plane augmentation

Surgeons often combine the two routes, placing the implant partly behind the breast and partly behind the muscle, to try to give the benefits of both — this is called a dual plane augmentation. If implants are inserted behind the muscle they are likely to move when the muscle contracts; this is called animation and is normal. Your surgeon will advise which placement is appropriate for you.

What the operation involves

Before and after photographs should be taken for your clinical record, to help you and your surgeon plan the operation and assess the result. Your face is not included and you will be asked for consent; if your surgeon wants to use the images for teaching or publication, they must ask for specific additional consent.

Breast enlargement surgery takes about one-and-a-half hours and is usually done under general anaesthetic. You may be able to go home the same day, but many patients spend one night in hospital. Postoperative pain should be well controlled: your chest may feel tight and your breasts and ribs may be tender. You will be mobile from day one and should be back to full exercise within six weeks, with one to two weeks off work recommended.

At first the breasts may look high and the skin tight; this tends to settle over the first six weeks, but the long-term result takes three to four months as a more natural shape emerges. Most patients are pleased with their surgery, though some find the new shape difficult to get used to. Scars are typically quite red for the first six weeks, change to purple over the next three months and then fade to white. Most patients form good-quality scars, but abnormal scarring can occur.

Implant techniques

Choosing the implant

Decision What the BAPRAS guide says Fill material The outer shell of all implants is made of silicone, and some have an additional polyurethane coating. The shell is filled with either silicone gel or saline. Implants have been used since the 1960s and the evidence suggests they are safe. Most patients are offered silicone gel-filled implants — they tend to feel the most natural, come in a range of shapes and are durable. Saline implants feel less natural, folds or ripples may be more visible, and they carry a risk of deflation. All other filler materials have been withdrawn for use in the UK. Ask your surgeon exactly which type and manufacturer will be used, and why. Size Implants are supplied by volume, and it is not possible to guarantee a cup size. Your surgeon assesses your chest wall, your existing breast tissue and how much skin is available, then indicates an appropriate range — your own view matters, because in most patients a range of sizes could be used. The larger the implant and the slimmer you are, the less natural the result; an implant out of proportion with your frame is more likely to cause complications such as drooping, stretching of the breast tissue and being more easily felt. Shape Implants are round or teardrop (anatomical). Round implants give more volume at the top of the breast and are as wide as they are tall. With teardrop implants the width and height can be chosen separately, giving more control of the eventual shape. Both come in varying degrees of projection; your surgeon discusses the best shape for your frame and desired outcome. Texture Surfaces are smooth or textured, in degrees from very fine to coarse; some implants have a foamy polyurethane coating. Texturing exists to reduce capsular contracture (evidence is strongest when the implant is placed in front of the muscle) and to reduce unwanted movement or rotation — so anatomical implants tend to be textured, while rotation of a round implant is not noticeable. A textured surface is linked to the uncommon BIA-ALCL; smooth implants currently have no such link. Smooth implants are almost always round, move within the breast and take several months to settle, and can slip sideways (lateral displacement) or drop below the crease (bottoming out). The choice is not straightforward — your surgeon should help decide. Silicone safety Implants are made from medical-grade silicone. Small quantities of silicone can be taken up by the body and found in breast tissue and sometimes the armpit lymph glands; occasionally the body forms inflammation and extra tissue around silicone deposits (silicone granuloma). If the shell fails, the gel often keeps its shape and the appearance may not change — ruptures are frequently discovered by accident while investigating another breast problem, though they can cause lumpiness or a change in shape. A confirmed rupture is treated by removal of the implants, with replacement possible at the same or a later operation. There is no association between breast augmentation and breast cancer.
The three most important decisions are implant size, shape and placement. Summarised from the BAPRAS implant techniques guide.

What complications can occur?

All operations are associated with risks. Serious complications are uncommon with breast augmentation, occurring in less than 1 in 100 operations. The figures below are those published in the BAPRAS guide.

Reported frequencies

Risks and how often they occur

Complication What the BAPRAS guide says Serious complications overall Uncommon — less than 1 in 100 breast augmentation operations. Bleeding (haematoma) Less than 1 in 100. Usually immediately after surgery, occasionally up to two weeks later. The breast becomes very swollen and tight and you return to theatre to have the blood removed and the bleeding stopped; the implant can be retained. Expect an extra night in hospital and more bruising than usual. Haematoma can increase the risk of later capsular contracture. Infection Less than 1 in 1000. Usually apparent over the first two or three weeks: swelling, tenderness, redness, wound discharge and a raised temperature. Antibiotics sometimes settle a mild infection, but most established infections around the implant require removal — a new implant cannot be inserted immediately, and three to six months should pass before re-insertion. Your package price should cover treatment of bleeding or infection. Capsular contracture The body forms a scar capsule around every implant; when it contracts the breast feels firmer, and may become round or tender. Noticeable firmness occurs in up to 1 in 10 of all breast augmentations, but most of those patients do not need revision surgery. Options include living with it (safe), releasing the capsule (capsulotomy) or removing it (capsulectomy) with a new implant — but scar tissue can return in at least half of people having secondary surgery. Capsular contracture is the most common reason for re-operation, and once it has happened it is likely to recur in 1 in 2 cases. Re-operation for any reason About 1% (1 in 100) per year — so after 10 years, about 10 in every 100 women will have had more surgery. Changes in sensation Some alteration of sensation is usual, most often numbness and oversensitivity of the nipples. Oversensitivity gradually settles, usually over several months. Around 1 in 5 patients have a reduction in nipple sensation; recovery can take up to 12 months, and 1 in 10 patients have some permanent numbness. Feeling or seeing the implant It is common to feel the implant, especially in slim patients or those with little breast tissue; this is an inevitable consequence of the operation and does not improve with time. Ripples or folds may become visible leaning forwards. Small amounts of your own fat can be injected (lipomodelling) to improve the situation in some patients. A marked crease can be a sign of capsular contracture. Implant failure (rupture) The shell can eventually fail. The American Core study (FDA update, 2011) found 1 in 10 implants had ruptured at 10 years, and it is estimated half of all breast implants may rupture by 15 years. Rupture is often contained within the body’s own capsule (silent rupture) and does not appear harmful; some patients notice a change in size, shape or consistency, a lump or swelling. A scan is usually performed and, if rupture is suggested, removal and exchange is advised. There is no universally agreed replacement schedule, and exchange before ten years is unusual — if you have noticed no change, no regular follow-up or scans are needed. Asymmetry Some asymmetry between breasts is normal and perfect symmetry is not an achievable goal; asymmetry may appear more pronounced after surgery. A larger implant in the smaller breast improves the volume difference only, not other differences such as nipple position. Implants can also settle differently in their pockets. Wide cleavage Implants are positioned centrally behind the nipples and will not improve a naturally wide cleavage — with sub-muscular placement the cleavage can even appear wider. Breasts and implants falling to the sides when you lie down is normal. Stretch marks and veins Stretch marks may develop after surgery, especially with larger implants, and veins may become more noticeable on the breast surface. Ptosis (sagging) Breasts will sag with time because augmented breasts are larger and heavier — the larger the implant, the more the breast will droop. They may also “bottom out”, meaning the implant sits lower, below the nipple rather than above it. Pregnancy and breastfeeding Implants do not interfere with the ability to breastfeed. There is some evidence that the amount of milk produced by some women with implants is reduced, and no evidence of increased illness in children of women with silicone gel implants. Pregnancy and breastfeeding may adversely affect the shape of the breast. Other reasons for re-operation Some patients choose re-augmentation with larger implants as time goes by; augmented breasts change shape with time, pregnancy and weight fluctuation; teardrop-shaped implants can rotate behind the breast (usually rotating back by themselves or being gently pushed back, but repeated rotation needs re-operation, especially with large implants correcting droop); and intermittent swelling around an implant may lead to scans, further tests or implant replacement. Who pays for future treatment Many of these long-term problems will not be covered by the package bought at the time of surgery, and you should not expect the NHS to provide future treatment. You must be prepared to pay for consultations, scans and further surgery if needed. Frequency figures are those published in the BAPRAS patient guide (updated 2021) and describe breast augmentation generally; individual risk depends on your anatomy, the technique used and your health.

Breast Implant Associated Anaplastic Large Cell Lymphoma (BIA-ALCL)

Since 2016, anaplastic large cell lymphoma in association with breast implants has been recognised by the World Health Organization. The risk is small. It is not a breast cancer but a type of cancer associated with the scar tissue (capsule) the body lays down around an implant. Cases have occurred between 2 and 28 years after implant insertion, with an average of 8 years, and it most likely presents as swelling around the implant (a seroma) causing an increase in breast size. It can usually be treated successfully by removing the implant and the surrounding capsule.

Because it is uncommon, international organisations share data on the condition. Most cases worldwide have occurred in women with textured breast implants, with higher numbers where the texture is coarser — so ask your surgeon for the most up-to-date recommendations. Breast implants continue to hold safety approval from government organisations such as the UK MHRA and the USA FDA and remain used in breast reconstruction after cancer worldwide.

Breast Implant Illness (BII)

Breast Implant Illness is a term used by some patients who experience symptoms they feel are connected to their silicone implants — tiredness, “brain fog”, joint aches, immune-related symptoms, sleep disturbance, depression, hormonal issues, headaches, hair loss, chills, rash and neurological issues. BII is not a medical diagnosis, there is no proven association with breast implants, and there is currently no scientific evidence confirming the proposed link or any diagnostic test for the condition; research continues. Some patients report improvement after implant removal, but not all do. The UK government guidance on BII symptoms and the FDA’s breast-implant risk pages are referenced in the guide.

Long-term consequences and alternatives

Mammography and screening

Implants push your natural breasts forward and do not make it harder to examine your breasts for lumps, but they do interfere with mammography: X-rays cannot pass through the implant, so some breast tissue is obscured. UK breast screening starts at age 50. If you are called for a screening mammogram you must tell the service you have implants — they may scan you at a different centre and take special views. The more of your total breast volume that is implant, the greater the problem, so a breast augmentation reduces the sensitivity of a future mammogram. Some patients pay privately for MRI screening instead to avoid these issues.

Myths and routine replacement

It is not unsafe to sunbathe or to fly in an aeroplane with implants — neither presents a problem. There is no need to massage breast implants, and this is not recommended with modern textured-surface implants. Routine replacement after a specified time is not recommended: replacement is advised only if you are unhappy with the appearance or have developed a problem.

Alternatives: lipofilling and breast uplift

The only alternative surgical technique to enlarge the breast is lipofilling (fat transfer): fat is removed by liposuction from the hips, thighs or tummy and injected into the breast area. Only a relatively small amount can be injected, so multiple operations are usually needed for a worthwhile effect; some fat is absorbed in the first weeks, but fat lasting beyond that brings permanent enlargement. It is more gradual than implant surgery and avoids an implant, but the surgeon cannot be sure how effective it will be and results are variable. Your own fat is the only substance that can be safely injected into the breast — other materials have been tried and withdrawn. Do not allow anyone to inject anything other than your own fat.

If you are bothered by droopiness but happy with your breast size, you do not need an augmentation — you may be offered a breast uplift (mastopexy), which lifts the nipple and breast tissue and tightens the skin, with incisions around the areola and possibly vertically and under the breast. Scarring is more obvious than with augmentation alone and no implant is used, but uplifted breasts tend to droop again with time and gravity, and often seem smaller afterwards despite little or no tissue removal. Mild droopiness can be improved by augmentation alone; where both volume loss and droopiness are a problem, augmentation and mastopexy may be discussed as one operation or in two stages — usually best done one at a time, though sometimes performed together after careful planning and counselling, with results that are not always entirely predictable in size and shape.

Breast Implant Registry

In the UK there is a national Breast and Cosmetic Implant Registry, administered by NHS Digital, recording every implant placed or removed with patient details, the organisation and the surgeon. Its main aim is to trace and inform affected patients if a failed implant is ever recalled, and to identify possible trends and complications relating to specific implants. Entry requires your consent, and you are strongly recommended to agree to the register if you have implant-based surgery — for your own protection and that of others.

Quick answers

Questions the BAPRAS guide answers

Can my surgeon guarantee a cup size?

No. Implants are supplied by volume and it is not possible to guarantee a cup size. Your surgeon assesses your chest wall, existing breast tissue and available skin, then indicates an appropriate range of sizes — in most patients a range could be used, and your own preference decides where in that range to sit.

Do breast implants have to be replaced after a set number of years?

No. There is no universally agreed replacement schedule, and exchange before ten years is unusual. If you have noticed no change you do not need regular follow-up or scans. You should still be prepared — personally and financially — for further surgery at some point, since about 1% of patients have a re-operation each year.

Is breast enlargement available on the NHS?

Cosmetically, no — you would consult and pay as a private patient. Exceptions include very uneven breasts, having no breasts, or treatment within a reconstruction care plan. If you have surgery abroad, only emergency complications will be treated by the NHS on your return.

What exactly is the cooling-off period?

At least two weeks between your decision and surgery, plus an offer of a second consultation beforehand. Nobody needs an urgent breast augmentation: if you are pressured to proceed or offered no cooling-off period, walk away.

Are silicone implants safe?

Implants are made from medical-grade silicone and have been used since the 1960s; the evidence suggests they are safe, and there is no association between breast augmentation and breast cancer. Rupture remains possible (about 1 in 10 at 10 years) and is usually treated by removal and exchange. Rare risks such as BIA-ALCL, and the symptoms described as breast implant illness, are covered above.

What is the alternative to an implant?

Lipofilling — transferring your own fat by liposuction and injection. It avoids an implant but usually needs several operations for a worthwhile effect, gives a gradual result and is variable. Your own fat is the only substance that should ever be injected into the breast.

Source and copyright

Summarised from the four-part BAPRAS patient guide to breast augmentation (updated 2021), edited by Lee Martin and Emma de Sousa. All original content remains the property of BAPRAS; this page is a condensed educational summary with links to the originals:

- Breast augmentation — introduction
- Implant techniques
- What complications can occur?
- Other information

BAPRAS is a UK professional association, so the guide describes UK practice and regulation. For published research, see the PMC research library ; for regulator guidance, the FDA and NHS libraries; for our own

treatment pages, breast augmentation and silicone implants .

Your journey starts with information

Take the guide’s questions into a consultation

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Breast Enlargement Research Library | PubMed Central || Quantum Health

Page: /research/breast-enlargement/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Evidence library

Breast Enlargement Research Library

The complete PubMed Central literature set for “breast enlargement” — searchable, with citations, abstracts and links to the original articles.

This library indexes every record returned by PubMed Central for the exact phrase “breast enlargement” , retrieved through the NCBI E-utilities on 2026-10-19. It is a reading aid for patients, students and clinicians: each entry links to the original record on pmc.ncbi.nlm.nih.gov , where the full text can be read in context.

How this library was built

The dataset is a complete capture of a single PubMed Central search rather than a hand-picked reading list. The query was the exact phrase "breast enlargement" , run against the PMC database with no date limit, subject filter or language filter. Records were retrieved in full, then enriched with citation metadata and abstracts from PubMed where a PMID was available.

The list is broad by design. It includes surgical augmentation, fat transfer, implants, gynaecomastia, neonatal and adolescent breast development, imaging, reconstruction and other uses of the phrase. Inclusion does not mean a study is relevant to you, high quality or current — and it is not an endorsement of any treatment. Abstract text belongs to the original authors and may differ from the published article.

Loading the research library…

Reading research responsibly

PubMed Central is an archive, not a recommendation service. A published paper can be small, observational, outdated or contradicted by later work. Before changing any treatment decision, read the full article, check its date and study design, and discuss it with a qualified clinician.

For independent systematic reviews and registered protocols on breast implants, wound drains and reconstruction, see the Cochrane evidence library . For regulator guidance on breast procedures and injectables, see the FDA breast enlargement information library and the NHS information library . For UK surgical-association patient guidance, see the BAAPS information library . For Australian Government health advice, see the healthdirect information library . For academic patient education on augmentation, implants and fat transfer, see the Cleveland Clinic information library . For U.S. consumer health information on the augmentation operation and recovery, see the MedlinePlus information library . For a peer-reviewed clinical review of the operation itself, see the NCBI Bookshelf (StatPearls) breast augmentation library . For a general-audience overview of breast enlargement, the methods cited and the ones listed as ineffective, see the Wikipedia information library . For professional-society patient information on breast augmentation from the American Society of Plastic Surgeons, see the ASPS information library . For our own guides, see the evidence guide and safety pages . PMC content is subject to the NLM copyright policy ; this site stores only citation metadata and abstracts. For professional-association patient guidance, see the BAPRAS patient guide .

Your journey starts with information

Want help interpreting the evidence for your situation?

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss

them with a qualified, registered healthcare professional who can assess you in person.

Cleveland Clinic Breast Enlargement Information | Augmentation, Implants & Fat Transfer | Breast Enlargement Guide

Page: /research/cleveland-clinic-breast-enlargement/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Clinical library

Cleveland Clinic Breast Enlargement Information

Breast enlargement information published by the Cleveland Clinic — breast augmentation, breast implant types, fat transfer, surgery, recovery and complications, extracted and linked to the original Health Library pages.

This library collects the Cleveland Clinic's public Health Library information on breast enlargement: breast augmentation, breast implants and fat transfer breast augmentation. Every entry links to the original page on my.clevelandclinic.org , so the wording, review dates and any subsequent updates can be checked at source.

What this library does and does not mean

Cleveland Clinic is an academic medical center that publishes patient education; it does not endorse any clinic, surgeon or treatment package, and nothing here is a recommendation to have a procedure. Individual suitability and outcomes require assessment by a qualified clinician.

Summaries and key points are condensed from Cleveland Clinic pages for readability. They are not a substitute for the original documents, which may have been updated since this library was compiled on 2026-10-20. Always read the current version on my.clevelandclinic.org and discuss it with a qualified clinician.

Coverage

What the Cleveland Clinic information covers

The library is grouped into three areas, from the augmentation procedure itself to implants and fat transfer.

The procedure

What breast augmentation is, the two main types, consultation, preparation, anaesthesia, incisions, implant placement and aftercare.

Recovery and risks

Time off work, recovery timelines of six to eight weeks, side effects, complications from capsular contracture to rupture, and when to call the doctor.

Safety concerns

BIA-ALCL and textured implants, cancer screening and mammograms with implants, breastfeeding effects and long-term implant monitoring.

Breast implants

Implant types, age limits for saline and silicone, natural look and feel, sizing, placement, incision options and how long implants last.

Fat transfer

Candidacy, the liposuction and injection technique, lifelong results, one-to-two-cup-size limits, risks and mammograms afterwards.

Decisions and costs

Questions to ask at consultation, why people choose augmentation, insurance coverage, reversibility and breastfeeding considerations.

All Cleveland Clinic pages in this library

Grouped by topic. Each entry gives a short summary, the main extracted points and a direct link to the full Cleveland Clinic page.

Breast augmentation

The Cleveland Clinic overview of breast augmentation: what it is, the two main types (implants and fat transfer), why people choose it, consultation, preparation, the operation itself and aftercare.

Breast Augmentation: What it is, Types, Surgery & Recovery

Treatment overview · Medically reviewed, last updated 2021-08-24

Cleveland Clinic's overview of breast augmentation, a common cosmetic surgery that uses breast implants or fat transfer to increase the size and shape of the breasts. It covers the implant types (saline, structured saline, silicone, form-stable, round, smooth, textured), fat transfer augmentation, why people choose the procedure, how common it is, the consultation and preparation, the steps of surgery (anaesthesia, incision, implant placement, closing), what happens afterwards, benefits, side effects and complications, cancer screening with implants, recovery time, when to call the doctor, insurance, reversibility and breastfeeding.

- Breast augmentation is a surgical procedure that increases the size and shape of your breasts using breast implants or fat transfer.
- Two main types: breast implants and fat transfer augmentation; certain implants are only FDA-approved for certain ages.
- Saline implants are filled with sterile salt water; if they break, your body absorbs the saline naturally.
- Silicone implants are filled with silicone gel; if they break the gel can stay in the shell or leak into the breast, and regular checks with your surgeon may be needed.
- Form-stable ("gummy bear") implants keep their shape even if the shell breaks, are made of thicker gel and need a longer incision.
- Round implants make breasts look fuller and don't change the look of the breast if they rotate; smooth implants feel softest and move most naturally.
- Textured implants encourage scar tissue to adhere so they move less, but breast implant-associated anaplastic large cell lymphoma (BIA-ALCL) occurs most frequently with textured surfaces.
- In fat transfer augmentation, liposuction takes fat from the belly, flanks, back or thighs and injects it into the breasts; it is usually for a relatively small increase in size.
- People choose augmentation to change size, correct asymmetry, restore size after pregnancy or weight loss, correct unevenness after other breast surgery, or to improve confidence.
- Around 300,000 people in the United States have breast augmentation surgery every year; it is the most popular type of cosmetic surgery.
- At consultation your surgeon reviews medications, allergies, smoking, prior surgeries and breast history; you should have realistic expectations and be in good mental and physical health.
- Preparation may include a blood test, adjusting medications, stopping smoking, avoiding aspirin and anti-inflammatory drugs, and stopping recreational drugs.
- Arrange a driver and someone to stay the first night; take at least three days off work, or at least three weeks for

a labour-intensive job.

- Surgery is done under general anaesthesia or IV sedation; incisions can be at the inframammary fold, around the areola (periareolar) or through the armpit (transaxillary).
- Implants are placed either under the breast tissue in front of the muscle or behind the pectoral muscle, depending on implant type and how much size is increased.
- Stitches are usually removed in about one week; no heavy lifting for at least four weeks and no sports for up to six weeks.
- Full recovery takes about six to eight weeks on average; recovery varies by augmentation type and overall health.
- Side effects include soreness, nipple sensitivity, bruising, bleeding and swelling; immediate complications include infection, wound healing issues and hematoma.
- Longer-term complications include capsular contracture (scar tissue), implant sagging, symmastia, seroma (fluid around the implant) and implant rupture or deflation.
- BIA-ALCL is a rare immune-system cancer reported with textured implants; symptoms such as size or shape changes, swelling or lumps may appear two to eight years after surgery.
- Breast implants are not guaranteed to last a lifetime; pregnancy, weight loss and menopause may change the appearance of augmented breasts.
- Implants can make mammograms harder to read; the American College of Radiology and related bodies recommend breast examinations at an ACR-accredited facility, which can take special views.
- Breast augmentation is usually considered elective cosmetic surgery, so insurance generally does not cover it; reconstruction after cancer may be covered.
- Yes, augmentation is reversible: implants can be removed, replaced or repositioned by a plastic surgeon.
- Augmentation can affect nerves and ducts involved in lactation; implants placed below the muscle affect milk production less than those above, and periareolar incisions are more likely to reduce milk production.

Read the original on my.clevelandclinic.org →

Breast implants

Cleveland Clinic information on breast implants themselves: who gets them, implant types, natural look and feel, sizing and placement, how long they last, advantages, risks and recovery.

Breast Implants: Types, Surgery, Recovery & Risks

Treatment overview · Medically reviewed, last updated 2021-08-31

Cleveland Clinic's guide to breast implants: artificial silicone shells filled with silicone gel or saline, inserted by plastic surgeons for augmentation or reconstruction. It covers candidacy, reasons for getting implants, how common they are, how long implants last, which implants feel and look most natural, age limits for saline and silicone implants, sizing and placement, the four incision types, the procedure, recovery timeline, follow-up schedule, advantages, risks and the questions to ask your provider.

- Breast implants are artificial devices (prostheses): silicone shells filled with silicone gel or saline, inserted by plastic surgeons.
- Candidates should be finished developing, in good physical health and have realistic expectations.
- People get implants to restore previous breast size after pregnancy, weight loss or aging, to correct asymmetry, to restore breasts after mastectomy, or to boost confidence.
- About 3.5 million people in the United States have breast implants; roughly 60% contain silicone gel; about 35 million people worldwide have implants.
- Breast augmentation is the procedure that makes breasts larger; breast implants are the devices used in many augmentations.
- Implants typically last about 10 years; the older they get, the more likely they are to rupture; signs include changes in size, shape, firmness or swelling, though rupture can also have no signs.

- Silicone implants tend to feel more natural than saline; smooth shells feel more natural and move more freely than thicker textured shells.
- The FDA approved highly cohesive silicone gel ("gummy bear") implants in 2012; teardrop-shaped implants usually come with a textured shell so they don't rotate.
- You must be at least 18 years old for saline-filled implants and the FDA recommends at least 22 years old for silicone-filled implants.
- Providers assess mental health and social factors, including body dysmorphic disorder concerns and whether a partner is pressuring the decision.
- Implant size is matched to body structure: larger implants suit broader shoulders or wider hips, while oversized implants on a small frame can cause back, neck and shoulder pain.
- Preoperative sizing lets you try sample implants under clothing in front of a mirror before choosing size and shell type.
- Two placement options: submuscular (beneath chest muscles) and subglandular (between chest muscles and breast tissue).
- Four incision types: periareolar (around the areola), inframammary (breast crease), transaxillary (armpit) and transumbilical (belly button).
- After surgery breasts are wrapped in a supportive bra or compression band; you need a driver and someone to care for you for the first 24 hours.
- Full recovery is on average six to eight weeks; the first several days are the most painful, with swelling, bruising and slight incision bleeding.
- Pain usually decreases after five to seven days; light daily activities and desk work can resume after one to two weeks, but avoid running, heavy lifting and arms above head.
- Return to work is typically between two and six weeks, depending on how physically demanding the job is.
- Advantages: implants are durable (about 10 years), implant surgery has a relatively low risk of complications, and implants can boost self-esteem.
- Risks include short- and long-term complications, the need for additional surgery (removal, replacement, scar-tissue removal or repositioning), BIA-ALCL and breastfeeding complications.
- Implants increase size but do not lift; a breast lift can be combined if sagging is a concern.
- Follow-up: bandages and possibly stitches removed after one to two weeks, then reviews at four to six weeks, three months and six months.
- Ask which implant type and size suit your body, how long implants last, what happens on rupture, what autologous breast augmentation is, and the full list of risks.

Read the original on my.clevelandclinic.org →

Fat transfer breast augmentation

Cleveland Clinic information on using your own fat (liposuctioned and reinjected) to enlarge the breasts: candidacy, the procedure, how long results last, benefits, downsides, recovery and mammograms afterwards.

Fat Transfer Breast Augmentation: Procedure, Results & Risks

Treatment overview · Medically reviewed, last updated 2022-08-18

Cleveland Clinic's guide to fat transfer breast augmentation: liposuction removes fat from areas such as the belly, hips, back, legs or arms, and the purified fat is injected into the breasts to increase cup size by one to two sizes. It explains how the procedure differs from implants and reconstruction, who is and is not a candidate, how the outpatient surgery is performed, that results are lifelong (settling over up to six months), the risks, benefits and cons, recovery, when to call the doctor, and the effect on breast cancer risk and mammograms.

- A fat transfer breast augmentation uses fat from your own body to enlarge your breasts: fat is suctioned from several areas and injected into the breasts.
- Breast augmentation is cosmetic surgery to increase breast size and enhance shape; the two types are breast

implants and fat transfer.

- Breast augmentation is for healthy people improving appearance and is not medically necessary; breast reconstruction after cancer always uses implants.
- Good candidates want slightly larger breasts without implants; people with little body fat, obesity, drastic weight fluctuations or who smoke may not be suitable.
- It is an outpatient procedure under general anaesthesia: fat is harvested by liposuction, purified in a centrifuge, then injected into different tissue layers of the breasts.
- Results are described as lifelong: fat cells settle over up to six months and remain for life, though extra fat is injected because some cells die after transfer.
- Risks include bleeding and bruising, fat cell death (necrosis) or fat migration, infection, and benign breast cysts or calcifications.
- Benefits over implants: a natural look and feel, the donor areas (belly, hips, back, legs or arms) become slimmer, and no implant-related risks such as rupture or capsular contracture.
- Cons: only one or two cup sizes of increase, breasts may still sag (a breast lift would be needed), and multiple procedures may be required.
- Recovery: chest wrapped with an elastic bandage and surgical bra, about a week off work, pain bruising and swelling for up to three weeks, swelling may last up to six months.
- Sleep on your back, take antibiotics and pain relievers as directed, and wear loose clothing to ease recovery.
- Call your provider for excessive bleeding, swelling or bruising, nausea and vomiting, severe pain, or signs of infection such as yellow discharge or fever.
- Fat transfer does not increase breast cancer risk and mammograms still work well afterwards, though it can cause microcalcifications that may need further tests such as ultrasound, MRI or needle biopsy.

Read the original on my.clevelandclinic.org →

Extracted data

Key facts from the Cleveland Clinic pages

The statements below summarise what Cleveland Clinic says about breast enlargement and related topics. The right-hand column links to the source page.

Topic What Cleveland Clinic says Source What breast augmentation is Breast augmentation is a common surgical procedure that increases the size and shape of your breasts using breast implants or fat transfer. Breast Augmentation: What it is, Types, Surgery & Recovery Two main types There are two main types of breast augmentation: breast implants and fat transfer augmentation, each with several options based on how you want your breasts to look and feel. Breast Augmentation: What it is, Types, Surgery & Recovery Saline implants Saline breast implants are filled with sterile salt water; if an implant breaks inside your breast, your body absorbs the saline and naturally gets rid of it. Breast Augmentation: What it is, Types, Surgery & Recovery Structured saline implants Structured saline implants are also filled with sterile saline but have an inner structure that helps the implant feel more natural. Breast Augmentation: What it is, Types, Surgery & Recovery Silicone implants Silicone breast implants are made of silicone gel; if the implant breaks the gel can stay within the shell or leak into the breast, and regular checks with your plastic surgeon may be required. Breast Augmentation: What it is, Types, Surgery & Recovery Form-stable implants Form-stable (gummy bear) implants keep their shape even if the shell breaks; they use a thicker silicone gel, are firmer than traditional implants and need a longer skin incision. Breast Augmentation: What it is, Types, Surgery & Recovery Round vs smooth vs textured Round implants make breasts look fuller and don't change the look of the breast if they rotate; smooth implants feel softest and move most naturally; textured implants adhere to scar tissue so they move less. Breast Augmentation: What it is, Types, Surgery & Recovery Textured implants and BIA-ALCL Breast implant-associated anaplastic large cell lymphoma (BIA-ALCL), though rare, occurs most frequently in people with breast implants that have textured surfaces. Breast Augmentation: What it is, Types, Surgery & Recovery Fat transfer augmentation In fat transfer augmentation, liposuction takes fat from the belly, flanks, back or thighs and injects it into the breasts; it is usually for people wanting a relatively small increase in breast size. Breast Augmentation: What it is, Types, Surgery & Recovery Reasons for augmentation People choose breast

augmentation to change breast appearance, correct asymmetry, restore size after pregnancy or weight loss, correct unevenness after other breast surgery, or improve confidence and self-esteem. Breast Augmentation: What it is, Types, Surgery & Recovery How common it is Breast augmentation is the most popular type of cosmetic surgery; around 300,000 people in the United States have the surgery every year. Breast Augmentation: What it is, Types, Surgery & Recovery Consultation questions At consultation you should ask whether the surgeon is board-certified (American Board of Plastic Surgery), how often they perform breast augmentations, whether you can see before-and-after pictures, whether implants or fat transfer suit you, and what happens if you are not satisfied with the results. Breast Augmentation: What it is, Types, Surgery & Recovery Preparation Preparation may include a blood test, adjusting medications, stopping smoking, avoiding aspirin and certain anti-inflammatory drugs (which increase bleeding), and stopping recreational drugs. Breast Augmentation: What it is, Types, Surgery & Recovery Time off work You need someone to drive you home and stay with you the first night; take at least three days off work, or at least three weeks if you have a labour-intensive job. Breast Augmentation: What it is, Types, Surgery & Recovery Anaesthesia and incisions Surgery is performed under general anaesthesia or IV sedation; incisions can be made at the inframammary fold (crease under the breast), around the areola (periareolar) or through the armpit (transaxillary). Breast Augmentation: What it is, Types, Surgery & Recovery Implant placement Implants are placed either under the breast tissue in front of the muscle or behind the pectoral muscle; placement depends on implant type and how much size you are increasing. Breast Augmentation: What it is, Types, Surgery & Recovery After surgery After surgery you are observed while waking up (usually about an hour), given pain medication and follow-up instructions, and sent home with a gauze bandage and possibly a surgical bra; drainage tubes are removed as instructed. Breast Augmentation: What it is, Types, Surgery & Recovery Recovery timeline Stitches are usually removed in about one week; no heavy lifting for at least four weeks; sports may take up to six weeks; full recovery averages six to eight weeks. Breast Augmentation: What it is, Types, Surgery & Recovery Side effects Side effects can include soreness and increased nipple sensitivity, bruising, bleeding and swelling. Breast Augmentation: What it is, Types, Surgery & Recovery Immediate complications Immediate complications, though rare, can include infection, wound healing issues and hematoma (blood collection in the surgical area). Breast Augmentation: What it is, Types, Surgery & Recovery Long-term complications Long-term complications can include capsular contracture (scar tissue formation), implant sagging, symmastia (implants drifting toward the middle of the chest), seroma (fluid around the implant) and implant rupture or deflation. Breast Augmentation: What it is, Types, Surgery & Recovery Implants are not lifelong Breast implants are not guaranteed to last your entire life; you may need another procedure to replace one or both implants, and pregnancy, weight loss and menopause may affect the result. Breast Augmentation: What it is, Types, Surgery & Recovery Cancer screening Implants can make self-examination and mammograms more difficult; the American College of Radiology, American Cancer Society, American Society of Breast Imaging and American Society of Plastic and Reconstructive Surgeons recommend breast examinations at an ACR-accredited facility capable of special views. Breast Augmentation: What it is, Types, Surgery & Recovery Insurance Breast augmentation is usually considered elective cosmetic surgery, so insurance generally does not cover the procedure or follow-up visits; reconstruction after breast cancer may be covered. Breast Augmentation: What it is, Types, Surgery & Recovery Reversibility Breast augmentation is reversible: a plastic surgeon can remove, replace or reposition implants through breast implant removal surgery. Breast Augmentation: What it is, Types, Surgery & Recovery Breastfeeding Augmentation can affect nerves and ducts involved in lactation; implants placed below the muscle affect milk production less than those above it, and periareolar incisions are more likely to reduce milk production. Breast Augmentation: What it is, Types, Surgery & Recovery When to call the doctor Contact your healthcare provider immediately if you develop fever, abnormal discharge such as pus from the incision, rupture of your stitches, or increasing enlargement or firmness of a breast with pain and pressure. Breast Augmentation: What it is, Types, Surgery & Recovery What implants are Breast implants are artificial silicone shells filled with silicone gel or saline, inserted by plastic surgeons to change the size or shape of the breasts. Breast Implants: Types, Surgery, Recovery & Risks Implant prevalence About 3.5 million people in the United States have breast implants (roughly 60% silicone gel), and around 35 million people worldwide. Breast Implants: Types, Surgery, Recovery & Risks Implant longevity New breast implants typically last about 10 years; the older they are, the more likely they are to rupture, sometimes without any signs such as changes in size, shape, firmness or swelling. Breast Implants: Types, Surgery, Recovery & Risks Age limits You have to be at least 18 years old for saline-filled breast implants, and the FDA recommends being at least 22 years old for silicone-filled implants. Breast Implants: Types, Surgery, Recovery & Risks Sizing Larger implants often suit broader shoulders or wider hips, while larger implants on a smaller frame can cause back, neck and shoulder pain; preoperative sizing lets you try samples under clothing before deciding. Breast Implants: Types, Surgery, Recovery & Risks Natural look

and feel Silicone implants tend to feel more natural than saline; submuscular placement means you only feel natural breast tissue on top of the implant, and smooth shells feel and move more naturally than textured shells. Breast Implants: Types, Surgery, Recovery & Risks Incision types Implant incisions can be periareolar (around the areola), inframammary (breast crease), transaxillary (armpit) or transumbilical (belly button). Breast Implants: Types, Surgery, Recovery & Risks Risks of implants Risks of breast implants include short- and long-term complications, additional surgery for removal, replacement, scar-tissue removal or repositioning, BIA-ALCL, and breastfeeding complications. Breast Implants: Types, Surgery, Recovery & Risks Implants do not lift Breast implants increase size but do not lift the breasts; if sagging is a concern, discuss combining a breast lift with implants. Breast Implants: Types, Surgery, Recovery & Risks Follow-up schedule After implant surgery, bandages and possibly stitches are removed at one to two weeks, with further reviews at four to six weeks, three months and six months. Breast Implants: Types, Surgery, Recovery & Risks Fat transfer basics Fat transfer breast augmentation suctions fat from areas such as the belly, hips, back, legs or arms and injects the purified cells into the breasts, so donor areas become slimmer while the breasts get bigger. Fat Transfer Breast Augmentation: Procedure, Results & Risks Fat transfer candidacy People with little body fat may not be good candidates because more fat must be removed than used; people with obesity, drastic weight fluctuations or who smoke may also be unsuitable. Fat Transfer Breast Augmentation: Procedure, Results & Risks Fat transfer procedure The procedure is an outpatient surgery under general anaesthesia: fat is harvested by liposuction, purified in a centrifuge, then injected into different tissue layers of the breasts. Fat Transfer Breast Augmentation: Procedure, Results & Risks Fat transfer results Results from a fat transfer breast augmentation are described as lifelong; final results can take up to six months to appear as fat cells settle, and extra fat is injected because some cells die after transfer. Fat Transfer Breast Augmentation: Procedure, Results & Risks Fat transfer limits Fat transfer increases breast size by only one or two cup sizes, does not fix sagging (a breast lift is needed for that), and may need multiple procedures to achieve the desired result. Fat Transfer Breast Augmentation: Procedure, Results & Risks Fat transfer risks Risks of fat transfer include bleeding and bruising, fat cell death (necrosis) or fat migrating to other parts of the body, infection, and benign breast cysts or calcifications. Fat Transfer Breast Augmentation: Procedure, Results & Risks Fat transfer recovery After fat transfer the chest is wrapped with an elastic bandage and surgical bra; plan about a week off work, with pain, bruising and swelling for up to three weeks and swelling that may persist up to six months. Fat Transfer Breast Augmentation: Procedure, Results & Risks Cancer risk and mammograms A fat transfer breast augmentation does not increase breast cancer risk and mammograms still work well afterwards, though it can cause microcalcifications that sometimes need ultrasound, MRI or needle biopsy to rule out cancer. Fat Transfer Breast Augmentation: Procedure, Results & Risks
Statements are paraphrased from Cleveland Clinic patient information. Pages are medically reviewed and updated periodically; verify against the linked my.clevelandclinic.org page before relying on any point.

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For regulator guidance, see the FDA information library and the NHS information library . For UK surgical-association patient guidance, see the BAAPS information library . For Australian Government health advice, see the healthdirect information library . For professional-society patient information on breast augmentation, see the ASPS information library . For published research, see the PMC research library .

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Your journey starts with information

Want to discuss what the Cleveland Clinic guidance means for

you?

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Cochrane Breast Enlargement Evidence: Systematic Reviews, Implants & Protocols | Breast Enlargement Guide

Page: /research/cochrane-breast-enlargement/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Systematic reviews

Cochrane Library Breast Enlargement Evidence

Cochrane systematic reviews and registered protocols relevant to breast enlargement and breast implants — wound drains, implant types, implants versus the patient's own tissue, mesh and fat transfer — summarised and linked to the original records on the Cochrane Library.

This library collects every Cochrane systematic review and registered review protocol that bears on breast enlargement, breast implants and implant-based reconstruction — 3 completed reviews and 4 protocols registered with, or withdrawn by, Cochrane. Each entry links to the full record on cochranelibrary.com , with its DOI and PubMed citation where one exists, so the wording, dates and any later updates can be checked at source.

What this library does and does not mean

Cochrane publishes independent systematic reviews that search for and appraise all available studies on a clinical question. A review here reports what the evidence showed when it was published; a protocol only states what a team intends to review — protocols contain no results, and one entry in this library was withdrawn before any review was written. Nothing here is a recommendation to have or avoid a procedure, and individual suitability always requires assessment by a qualified clinician.

Summaries and key points are condensed from the Cochrane abstracts and record pages for readability. They are not a substitute for the full reviews, which may have been updated since this library was compiled on 20 October 2026. Always read the current version on cochranelibrary.com.

Coverage

What the Cochrane evidence covers

The library is grouped into four areas, from the augmentation question itself to the reconstructive surgery evidence and the protocols still in progress.

Completed reviews

3 completed Cochrane Reviews: wound drains after breast surgery, different implant types, and implants versus autologous tissue flaps after mastectomy.

Augmentation directly

What the reviews say about breast augmentation itself — the drain trials, and why capsular-contracture research (implant insertion funnels) is still only a registered protocol.

Implant type evidence

Five small randomised trials (202 participants) comparing surfaces, fillers and shapes — and the review’s

conclusion that implant choice cannot yet be based on randomised evidence.

Reconstruction evidence

Quality of life, satisfaction and complications with implants versus the patient's own tissue across 35 non-randomised studies and 57,555 participants.

Protocols in progress

Registered protocols on insertion funnels, prepectoral mesh and acellular dermal matrices — what Cochrane is currently examining, with no results yet.

Evidence gaps

Where Cochrane has no review at all: cosmetic augmentation as a procedure, fat transfer for enlargement, breast uplift, fillers and non-surgical options.

All Cochrane records in this library

Grouped by topic. Each entry gives the record type, publication date, a short summary, the main findings or planned methods, and a direct link to the full record, DOI and PubMed citation.

Augmentation and implant technique

Cochrane records that address breast enlargement directly — wound drains after augmentation surgery and the implant insertion techniques used to prevent capsular contracture.

Wound drainage after plastic and reconstructive surgery of the breast

Cochrane Review · Published 21 October 2015

Whether surgical wound drains should be placed after breast augmentation, reduction or reconstruction. Three randomised trials (306 women, 505 breasts) were found out of 190 screened studies — all of them in breast reduction — comparing drains with no drains after surgery.

- Across the three trials, drains produced no statistically significant improvement in infection, other wound complications, pain, bruising, seroma, haematoma or reoperation.
- Women who were not drained left hospital a mean 0.77 days sooner (95% CI 0.40 to 1.14 days sooner; low-certainty evidence) — the only significant result.
- No trials of drains after breast augmentation or breast reconstruction were identified, so the findings apply to breast reduction only.
- Cochrane concludes that the limited evidence shows no significant benefit of routine post-operative drains, and that augmentation and reconstruction still require investigation.

Read the record on cochranelibrary.com → · DOI 10.1002/14651858.CD007258.pub3 · PMID 26487173

Breast implant insertion funnels for preventing capsular contracture

Cochrane Protocol · Review in progress · Published 30 March 2026

Registered protocol for a review that will assess the effects of breast implant insertion funnels — devices used to guide the implant into place while minimising contact with skin — on capsular contracture rates in women having breast reconstruction or breast augmentation.

- This is a protocol, not a completed review: no results are available yet.
- The primary outcome planned is capsular contracture, alongside other complications, with both randomised and non-randomised studies eligible.
- The protocol anticipates few, if any, randomised trials of funnels, because capsular contracture often appears years after surgery and has many causes.

Read the record on cochranelibrary.com → · DOI 10.1002/14651858.CD016344 · PMID 41909996

Implant types and materials

Cochrane reviews comparing implant surfaces, fillers and shapes, and what the randomised evidence does and does not establish about choosing between them.

Different types of implants for reconstructive breast surgery

Cochrane Review · Published 16 May 2016

Whether different breast implant types — textured or smooth, silicone or saline, anatomical or round, variable or fixed volume — change capsular contracture, complications, satisfaction and quality of life after reconstructive breast surgery. Five randomised trials with 202 participants were found.

- Every comparison was rated very low quality evidence: the trials were small, at unclear or high risk of bias, and too different from each other to pool.
- In one trial of 60 women, saline implants had less severe capsular contracture and higher satisfaction than silicone implants, but reinterventions were significantly more frequent with saline.
- No comparison reached statistical significance for textured versus smooth silicone implants (1 study, 20 women) or anatomical versus round implants (1 study, 36 women).
- No trials compared fifth-generation silicone implants or different manufacturers; the review concludes that surgeons still cannot counsel women on implant choice on the basis of randomised evidence.

Read the record on cochranelibrary.com → · DOI 10.1002/14651858.CD010895.pub2 · PMID 27182693

Implant-based reconstruction

Reviews and registered protocols on reconstruction after mastectomy: implants versus the patient's own tissue, and the meshes and dermal matrices used to support implants.

Implants versus autologous tissue flaps for breast reconstruction following mastectomy

Cochrane Review · Published 31 October 2024

Implant-based versus own-tissue (flap) reconstruction after mastectomy — quality of life, satisfaction and short- and long-term complications. Thirty-five non-randomised studies with 57,555 participants met the criteria; no randomised trials were found.

- Implants may reduce psychosocial well-being (mean difference -4.26 on a 0–100 scale; low-certainty evidence) and sexual well-being (-6.63; low-certainty) versus autologous flaps on BREAST-Q measures.
- Women may be less satisfied with the reconstruction with implants (-5.96; low-certainty) and with the breast itself (-8.17; very low-certainty).
- Long-term complications may be more frequent with implants (risk ratio 1.56, 95% CI 1.09 to 2.22; very low-certainty), while implant surgery was about 125 minutes shorter (very low-certainty).
- All evidence came from non-randomised studies (26 at serious overall risk of bias); the review found no superior technique for every woman and called for randomised trials.

Read the record on cochranelibrary.com → · DOI 10.1002/14651858.CD013821.pub2 · PMID 39479986

Mesh for immediate implant-based prepectoral breast reconstruction following mastectomy

Cochrane Protocol · Review in progress · Published 17 June 2025

Registered protocol for a review of biological or synthetic mesh used to support a prepectoral implant (placed above the muscle) in immediate breast reconstruction after mastectomy.

- This is a protocol, not a completed review: no results have been published.
- It plans to compare mesh versus no mesh, and biological versus synthetic mesh, for complications, patient-reported outcomes and reoperation after prepectoral reconstruction.
- Prepectoral placement is one of the placement options also discussed in breast augmentation, so the review will

be relevant to implant positioning debate generally.

Read the record on cochranelibrary.com → · DOI 10.1002/14651858.CD015961 · PMID 40525534

Acellular dermal matrices for breast reconstruction surgery

Cochrane Protocol · Review in progress · Published 2015

Registered protocol for a review of acellular dermal matrices (biological meshes derived from human or animal skin) used around implants in immediate prosthetic breast reconstruction after cancer or risk-reducing surgery.

- This protocol was registered in 2015; no completed review has been published under this title.
- It plans to compare reconstruction with versus without acellular dermal matrix for seroma, infection, skin flap necrosis, haematoma, reconstruction failure and implant removal.
- The protocol notes that some studies suggest dermal matrix may raise complication rates, which the review is intended to test.

Read the record on cochranelibrary.com → · DOI 10.1002/14651858.CD011966

Fat transfer

The Cochrane record on fat grafting — withdrawn before any review was completed — and the evidence gap it leaves.

Autologous fat grafting for breast reconstruction

Cochrane Protocol · Withdrawn · Published 29 June 2021

Protocol for a review of autologous fat grafting — transferring the patient's own fat into the breast — for breast reconstruction after cancer surgery. Cochrane withdrew the protocol because the authors were no longer able to complete the review.

- Cochrane withdrew this protocol: "the authors are no longer able to complete this review", so no Cochrane systematic review of fat grafting exists under this title.
- The protocol itself was published on 29 June 2021 and remains readable as a record of the intended methods.
- No Cochrane review has examined fat transfer for cosmetic breast enlargement either.

Read the record on cochranelibrary.com → · DOI 10.1002/14651858.CD010793.pub2

Extracted data

Key findings from the Cochrane records

The statements below summarise what the Cochrane reviews found, and what the protocols and evidence gaps mean for breast enlargement. The right-hand column links to the source record.

Topic What the Cochrane evidence shows Source Wound drains Randomised evidence does not support routine drains after breast surgery: in three trials (306 women) drains did not reduce complications, and hospital stay was about 0.8 days shorter without them. Every trial was in breast reduction — none in augmentation. Wound drainage after plastic and reconstructive surgery of the breast Implant type No randomised trial of usable quality establishes one implant type as better than another. All five trials (202 participants) were rated very low quality evidence, and neither textured-versus-smooth nor anatomical-versus-round comparisons reached significance. Different types of implants for reconstructive breast surgery Implants versus own tissue Across 35 non-randomised studies (57,555 participants), implants may mean lower psychosocial and sexual well-being and less satisfaction than autologous flaps, but about 125 minutes shorter surgery — with low to very low certainty throughout. Implants versus autologous tissue flaps for breast reconstruction following mastectomy Capsular contracture Whether implant insertion funnels reduce capsular contracture is being assessed in a review registered in 2026. No results exist yet, and the protocol expects few randomised trials to be found. Breast implant insertion funnels for preventing capsular contracture Mesh and dermal matrix Two registered protocols — mesh for prepectoral support and acellular dermal matrices — still have no completed review, leaving implant-support techniques without a Cochrane synthesis. Mesh for immediate implant-based

prepectoral breast reconstruction following mastectomy Fat transfer Cochrane has never published a review of fat transfer for breast enlargement. The protocol on fat grafting for reconstruction was withdrawn in 2021 because the authors could not complete it. Autologous fat grafting for breast reconstruction
Findings are paraphrased from Cochrane review abstracts (retrieved via PubMed, October 2026) and are stated with the certainty ratings Cochrane applied to them. Protocols report planned methods only. Verify each point against the linked record on cochranelibrary.com before relying on it.

Where the Cochrane evidence runs out

Cochrane has published no systematic review of cosmetic breast augmentation as a procedure — no review of augmentation techniques, implant placement planes, satisfaction after cosmetic surgery, or long-term outcomes in women who had implants for enlargement rather than reconstruction. The completed reviews here were written for breast reduction and post-mastectomy reconstruction; their findings are evidence about those settings, not direct evidence about augmentation for cosmetic purposes.

There is also no Cochrane review of fat transfer or autologous fat grafting for cosmetic breast enlargement (the reconstruction protocol was withdrawn in 2021), none of silicone gel implants used for cosmetic purposes, none of breast uplift (mastopexy), none of gynaecomastia surgery in men, and none of the non-surgical options discussed elsewhere on this site — fillers, platelet-rich plasma, threads or biostimulators. Absence of a Cochrane review does not mean a treatment works or does not work; it means the question has not been systematically synthesised at this level, and other evidence must be weighed instead.

Search the Cochrane Library yourself

Cochrane's own search can be run directly: search the Cochrane Library for breast augmentation . Reviews carry identifiers of the form CD012345 (with .pubN suffixes for updated editions); the Cochrane Central Register of Controlled Trials (CENTRAL) inside the Library also indexes thousands of individual trials on breast implants and breast surgery that no review has yet summarised.

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The structured dataset behind this page is available as JSON . It contains the record list, categories, summaries, key points and extracted findings, with DOI, PMID and source URLs.

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Cochrane reviews and protocols are published in the Cochrane Database of Systematic Reviews in the Cochrane Library and remain the property of Cochrane and the respective authors, with abstracts distributed by PubMed. This library stores only our own summaries, paraphrased key points and links to the publicly available records; full abstracts and review texts are read on cochranelibrary.com.

For the wider published literature, see the PMC research library . For regulator guidance, see the FDA information library and the NHS information library . For academic patient education, see the Cleveland Clinic information library . For professional-society patient information on breast augmentation, see the ASPS information library .

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Want to discuss what the Cochrane evidence means for you?

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

FDA Breast Enlargement Information | Breast Implants, Fillers & Safety || Quantum Health

Page: /research/fda-breast-enlargement/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Regulator library

FDA Breast Enlargement Information

Breast enlargement information published by the U.S. Food and Drug Administration — breast implants, BIA-ALCL, dermal fillers and injectable silicone, extracted and linked to the original fda.gov pages.

This library collects the FDA's public information on breast enlargement and the products used for it. Every entry links to the original page on fda.gov , so the wording, dates and any subsequent updates can be checked at source.

What this library does and does not mean

The FDA regulates medical devices such as breast implants and dermal fillers. It does not approve breast enlargement as a procedure, and it does not endorse any clinic, surgeon or treatment package. A document appearing here is not a recommendation to have a treatment.

Summaries and key points are condensed from FDA pages for readability. They are not a substitute for the original documents, which may have been updated since this library was compiled on 2026-10-19. Always read the current version on fda.gov and discuss it with a qualified clinician.

Coverage

What the FDA information covers

The library is grouped into five areas, from implant regulation and safety to fillers, cancers and patient reporting.

Implant regulation

Approved implant types, age indications, labeling requirements and restrictions on sale and distribution.

Safety and complications

Capsular contracture, rupture, reoperation, monitoring, breast implant illness and long-term follow-up.

Cancers

BIA-ALCL, squamous cell carcinoma and other lymphomas reported in the capsule around breast implants.

Fillers and silicone

Approved facial uses, the FDA position on fillers for breast enlargement, and warnings about injectable silicone.

Consumer guidance

Decision checklists, patient labeling and how to report adverse events or suspected illegal products.

All FDA documents in this library

Grouped by topic. Each entry gives a short summary, the main extracted points and a direct link to the full FDA page.

Breast implants — regulation and approval

How the FDA regulates saline and silicone gel-filled breast implants, which uses are approved, labeling requirements and restrictions on sale.

Breast Implants

Regulatory hub · Content current as of 2023-12-15

The FDA's central breast implant resource. It explains that implants are medical devices placed under breast tissue or chest muscle to increase breast size (augmentation) or to replace tissue removed by cancer or trauma, and links to approvals, labeling, risks, safety communications and post-approval studies.

- Two implant types are approved for sale in the United States: saline-filled and silicone gel-filled. Both have a silicone outer shell.
- Implants vary in size, shell thickness, surface texture and shape.
- The FDA provides this information to support, not replace, the discussion between a patient and a physician.
- Patients are encouraged to report adverse events through MedWatch.

Read the original on fda.gov →

Types of Breast Implants

Regulatory page · Content current as of 2019-10-23

Describes saline-filled and silicone gel-filled implants, their approved age groups, and the role of breast tissue expanders in reconstruction.

- Saline-filled implants are approved for breast augmentation in women aged 18 or older and for reconstruction at any age.
- Silicone gel-filled implants are approved for breast augmentation in women aged 22 or older and for reconstruction at any age.
- Breast implants are not lifetime devices; the longer they are in place, the more likely removal or replacement becomes.
- The most frequent complications are capsular contracture, reoperation and implant removal, with or without replacement.
- MRI remains the most effective method for detecting silent rupture; ultrasound is an acceptable alternative for screening asymptomatic patients.

Read the original on fda.gov →

Breast Implants — Certain Labeling Recommendations to Improve Patient Communication

FDA guidance (2020) · Content current as of 2020-09-29

Final guidance recommending content and format for breast implant labeling, including a boxed warning, patient decision checklist, device description and rupture screening recommendations.

- Labeling should include a boxed warning and a patient decision checklist.
- Materials and device descriptions should list the types and quantities of chemicals and heavy metals found in or released by the implant.
- The guidance underpins the new labeling approved on 27 October 2021.
- The recommendations supplement the FDA's Saline, Silicone Gel, and Alternative Breast Implants guidance.

Read the original on fda.gov →

Saline, Silicone Gel, and Alternative Breast Implants

FDA guidance (2020) · Content current as of 2020-09-29

Guidance covering the information manufacturers should provide for saline, silicone gel and alternative breast

implants, superseding the 2006 version.

- Applies to saline, silicone gel and alternative breast implants.
- Supersedes the previous version dated 17 November 2006.
- Should be read alongside the labeling recommendations guidance.

Read the original on fda.gov →

FDA Strengthens Safety Requirements and Updates Study Results for Breast Implants

Press announcement · Content current as of 2021-10-27

Announcement of orders restricting the sale and distribution of breast implants and approval of new labeling with a boxed warning, patient decision checklist, rupture screening recommendations and a patient device card.

- Sale and distribution are restricted to providers and facilities that give patients the approved risk information.
- The patient decision checklist must be reviewed, initialled and signed by the patient and signed by the implanting physician.
- New labeling includes a device description listing specific materials in the device.
- The FDA also released updated information on manufacturer post-approval studies.

Read the original on fda.gov →

Breast implants — safety and complications

Known risks, complications, rupture screening, revision surgery, monitoring and long-term follow-up for breast implants.

Risks and Complications of Breast Implants

Regulatory page · Content current as of 2023-12-15

A detailed list of local complications and adverse outcomes reported in at least one percent of breast implant patients, plus sections on reoperation, removal, capsular contracture, rupture, BIA-ALCL, systemic symptoms, breastfeeding and effects on children.

- Listed complications include asymmetry, breast pain, capsular contracture, deflation, delayed wound healing, extrusion, hematoma, infection, malposition, necrosis, sensation changes, rupture, seroma, wrinkling and unsatisfactory size or style.
- Capsular contracture is graded I–IV using the Baker scale; grades III and IV are considered severe and may need reoperation.
- Silicone gel ruptures are often silent; MRI is the most effective detection method and ultrasound is an acceptable alternative for asymptomatic screening.
- The FDA has not detected an association between silicone gel-filled implants and connective tissue disease, breast cancer or reproductive problems, but notes that larger and longer studies would be needed to rule these out.
- Some patients report systemic symptoms grouped under the term breast implant illness (BII); causes are poorly understood.
- Some women can breastfeed after augmentation and some cannot; implant removal without replacement can leave dimpling, concavity, puckering or sagging.

Read the original on fda.gov →

Things to Consider Before Getting Breast Implants

Patient decision guide · Content current as of 2023-03-08

The FDA's pre-surgery checklist covering informed consent, reoperation, monitoring, BIA-ALCL, BII, surgical mesh and the information patients should discuss with their surgeon.

- Patients should assume additional surgery may be needed because implants are not lifetime devices and

complications can occur.

- Implant removal or replacement may not be covered by insurance, even when a complication occurs.
- The most common local complications are capsular contracture, reoperation and implant removal; others include rupture or deflation, wrinkling, asymmetry, scarring, pain and infection.
- BIA-ALCL is a cancer of the immune system, not breast cancer; it is treatable but can be fatal, and treatment usually involves removing the implant and capsule.
- Surgical mesh used in breast implant surgery has not been approved by the FDA for that use.
- Silicone implants may require regular MRI or ultrasound screening for silent rupture, which may not be covered by insurance.

Read the original on fda.gov →

Breast Implant Surgery

Regulatory page · Content current as of 2023-08-03

Explains where breast implant surgery can be performed, the FDA position on surgical mesh, and questions patients should ask their surgeon.

- Breast implant surgery can be performed in a hospital or a surgery centre.
- The FDA has not cleared or approved any surgical mesh for use with breast implants or in breast reconstruction.
- Patients are advised to discuss the benefits and risks of mesh with their health care provider.

Read the original on fda.gov →

Breast Implant Postmarket Safety Information

Regulatory page · Content current as of 2025-06-02

Describes the FDA's ongoing postmarket review of breast implant safety, including medical device reports, BIA-ALCL, systemic symptoms and manufacturer post-approval studies.

- The FDA continually evaluates postmarket safety and manufacturer compliance with approval conditions.
- Review includes medical device reports for BIA-ALCL and systemic symptoms in women with breast implants.
- Ongoing post-approval studies are listed by manufacturer and application number.
- In July 2019 Allergan voluntarily recalled Natrelle BIOCELL textured breast implants and tissue expanders.

Read the original on fda.gov →

BIA-ALCL and other cancers

Breast implant-associated anaplastic large cell lymphoma (BIA-ALCL), squamous cell carcinoma and other lymphomas reported in the capsule around implants.

Questions and Answers about Breast Implant-Associated Anaplastic Large Cell Lymphoma (BIA-ALCL)

Patient and clinician Q&A · Content current as of 2019-10-23

FDA answers on BIA-ALCL: what it is, who is at risk, symptoms, diagnosis, treatment and what patients and clinicians should do.

- BIA-ALCL is a T-cell lymphoma of the immune system, not breast cancer.
- It is usually found in the fluid or scar tissue near the implant and may spread in some cases.
- Risk is higher with textured-surface implants than with smooth-surface implants.
- Main symptoms are persistent swelling, a mass or pain around the implant, often years after surgery.
- Most patients are treated successfully with surgery to remove the implant and surrounding capsule; some also need chemotherapy or radiation.
- The FDA does not recommend routine removal of implants in patients without symptoms.

Read the original on fda.gov →

UPDATE: Reports of Squamous Cell Carcinoma (SCC) in the Capsule Around Breast Implants

Safety communication · Content current as of 2023-03-08

Updated safety communication on reports of squamous cell carcinoma and various lymphomas in the scar capsule around breast implants.

- The communication updates a September 2022 safety communication.
- The lymphomas described are not the same as BIA-ALCL.
- The FDA believes cases of SCC and lymphomas in the capsule may be rare.
- Health care providers should discuss these risks with women who have or are considering breast implants.
- Patients and providers are asked to report cancers around breast implants to the FDA.

Read the original on fda.gov →

Dermal fillers and injectable silicone

Approved and unapproved uses of dermal fillers, the FDA position on fillers for breast enlargement, and warnings about injectable silicone.

Dermal Fillers (Soft Tissue Fillers)

Regulatory page · Content current as of 2023-07-06

Describes approved facial and hand uses of dermal fillers, the risks, and the FDA's position on unapproved body uses including breast enlargement.

- Dermal fillers are approved for specific facial uses, lip and cheek augmentation, back-of-hand augmentation, HIV lipoatrophy and contour deficiencies.
- The FDA recommends against using dermal fillers or any injectable filler to increase breast size (breast augmentation) or for other body contouring.
- These uses are not approved by the FDA, and the risks of unapproved uses or unapproved products are not known.
- Injectable silicone is not approved for any aesthetic procedure and can cause long-term pain, infection, scarring, embolism, stroke and death.
- Needle-free devices are not approved for injecting dermal fillers, and no dermal filler is approved for over-the-counter use.
- Removal of filler may require injections, surgery or other interventions, and it may be difficult or impossible to remove permanent fillers.

Read the original on fda.gov →

FDA-Approved Dermal Fillers

Regulatory list

The FDA's list of approved dermal fillers and their approved indications, limited to facial tissue and the hands.

- The list covers approved facial and hand products.
- The FDA has not approved liquid silicone or silicone gel for injection.
- Approved indications vary by product; not every product is reviewed for every body site.

Read the original on fda.gov →

FDA Warns About Illegal Use of Injectable Silicone for Body Contouring and Associated Health Risks

Press announcement and safety communication · Content current as of 2017-11-13

Warning about injectable silicone and products falsely marketed as FDA-approved fillers to enlarge the buttocks, breasts and other body parts.

- Injectable silicone used for body contouring is not FDA-approved.
- Serious side effects include ongoing pain, scarring, tissue death, permanent disfigurement, embolism, stroke, infection and death.
- Complications may occur immediately or weeks, months or years later.
- Silicone injections for body contouring are often performed by unlicensed providers in non-clinical settings.
- Injectable silicone differs from the silicone contained inside approved breast implants, because the implant shell keeps the gel from migrating.

Read the original on fda.gov →

Consumer guidance and reporting

Patient-facing advice, decision checklists and how to report adverse events or suspected illegal products.

What to Know About Breast Implants

Consumer update · Content current as of 2025-01-31

Patient-facing overview of implant types, risks, labeling, long-term monitoring, BIA-ALCL and communication with a surgeon.

- The FDA has approved implants for augmentation, reconstruction after cancer or trauma, correction of developmental defects, and revision of previous surgery.
- Risks include additional surgery, BIA-ALCL, systemic symptoms (BII), capsular contracture, breast pain, rupture, deflation, silent rupture and infection.
- Breast implants are not lifetime devices and their lifespan cannot be predicted.
- Mandatory labeling includes a patient decision checklist, boxed warning and patient device card.
- Patients with silicone implants are advised to have regular screening for silent rupture and to tell mammography facilities that they have implants.
- Injectable silicone is not approved for breast augmentation, reconstruction or body contouring.

Read the original on fda.gov →

MedWatch: FDA Safety Information and Adverse Event Reporting Program

Reporting program

How patients and clinicians report injuries, adverse events or product problems related to breast implants, fillers and other FDA-regulated products.

- Voluntary reports can be filed online or by phone at 1-800-FDA-1088.
- Reports help the FDA monitor approved products after they are sold.
- Useful details include device name, manufacturer and a description of the event and any interventions.

Read the original on fda.gov →

Extracted data

Key facts from the FDA pages

The statements below summarise the FDA position on breast enlargement and related products. The right-hand column links to the source document.

Topic What the FDA says Source Implants are devices, not procedures The FDA regulates breast implants as medical devices and reviews their safety and effectiveness; it does not approve breast enlargement procedures

or clinics. Breast Implants Approved implant types Only saline-filled and silicone gel-filled breast implants are approved for sale in the United States. Both have a silicone outer shell. Breast Implants Age indications for augmentation Saline-filled implants are approved for breast augmentation from age 18; silicone gel-filled implants are approved for augmentation from age 22. Both are approved for reconstruction at any age. Types of Breast Implants Implants are not lifetime devices The longer implants remain in place, the more likely it is that they will need to be removed or replaced, and patients should assume further surgery may be needed. Things to Consider Before Getting Breast Implants Most frequent complications The most common local complications are capsular contracture, reoperation and implant removal; others include rupture or deflation, wrinkling, asymmetry, scarring, pain and infection. Things to Consider Before Getting Breast Implants Silent rupture Most silicone gel implant ruptures have no symptoms. MRI is the most effective detection method, and ultrasound is acceptable for screening asymptomatic patients. Risks and Complications of Breast Implants BIA-ALCL BIA-ALCL is a T-cell lymphoma of the immune system, not breast cancer. It is more common with textured implants, often presents as persistent swelling, a mass or pain, and is usually treated by removing the implant and capsule. Questions and Answers about Breast Implant-Associated Anaplastic Large Cell Lymphoma (BIA-ALCL) SCC and other lymphomas The FDA has received reports of squamous cell carcinoma and various lymphomas in the scar capsule around breast implants. These are not the same as BIA-ALCL and are believed to be rare. UPDATE: Reports of Squamous Cell Carcinoma (SCC) in the Capsule Around Breast Implants Breast implant illness Some patients report systemic symptoms such as fatigue, memory loss, rash, brain fog and joint pain, collectively called breast implant illness. The causes are poorly understood and research is ongoing. Risks and Complications of Breast Implants Fillers are not approved for breast enlargement The FDA recommends against using dermal fillers or any injectable filler to increase breast size. The use is not approved, and the risks of unapproved uses and products are not known. Dermal Fillers (Soft Tissue Fillers) Injectable silicone Injectable silicone is not approved for any aesthetic procedure. It can cause chronic pain, infection, scarring, permanent disfigurement, embolism, stroke and death. Dermal Fillers (Soft Tissue Fillers) Filler removal Removing or reducing filler may require injections, surgery or other interventions, and permanent or non-hyaluronic-acid fillers may be difficult or impossible to remove. Dermal Fillers (Soft Tissue Fillers) Labeling and consent Since 2021, FDA-approved breast implants carry a boxed warning, a patient decision checklist and a patient device card, and their sale is restricted to providers who give patients this information. FDA Strengthens Safety Requirements and Updates Study Results for Breast Implants Surgical mesh The FDA has not cleared or approved any surgical mesh for use with breast implants or in breast reconstruction. Breast Implant Surgery Reporting problems Patients and clinicians can report adverse events or product problems to the FDA through MedWatch online or by phone at 1-800-FDA-1088. MedWatch: FDA Safety Information and Adverse Event Reporting Program
Statements are paraphrased. Regulatory positions can change; verify against the linked FDA page before relying on any point.

Regulatory scope and jurisdiction

FDA statements describe the position of the U.S. regulator. Approvals, warnings and labeling requirements can differ in other countries, and the FDA does not regulate where a procedure is performed or by whom outside its jurisdiction. Local registration, advertising rules and clinical standards still apply.

For published research rather than regulator guidance, see the PMC research library . For UK patient information, see the NHS information library and the BAAPS information library . For Australian Government health advice, see the healthdirect information library . For academic patient education on augmentation and implants, see the Cleveland Clinic information library . For professional-society patient information on breast augmentation, see the ASPS information library . For U.S. consumer health information on the operation and recovery, see the MedlinePlus information library . For an evidence overview of non-surgical options, see the evidence guide . For professional-association patient guidance, see the BAPRAS patient guide .

Download and reuse

The structured dataset behind this page is available as JSON . It contains the document list, categories, summaries, key points and extracted facts, with source URLs. FDA website content is subject to the FDA website policies ; this library stores only summaries and links.

Your journey starts with information

Want to discuss what the FDA guidance means for you?

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

healthdirect Breast Augmentation Information | Breast Implants, Recovery & Risks | Breast Enlargement Guide

Page: /research/healthdirect-breast-enlargement/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Government library

healthdirect Breast Augmentation Information

Breast augmentation (breast implants) information published by healthdirect — what the procedure involves, benefits and considerations, preparation, recovery, complications, alternatives and Australian support resources, extracted and linked to the original page.

This library collects healthdirect's public patient information on breast augmentation (breast implants) and the Australian resources it links to. Every entry links to the original page on healthdirect.gov.au or to the Australian body named in the entry, so the wording, review dates and any subsequent updates can be checked at source.

What this library does and does not mean

healthdirect is a free Australian Government health information service, not a regulator or a clinic. Its pages describe breast augmentation as it is practised and paid for in Australia, what the operation involves and what can go wrong. Nothing here is a recommendation to have a procedure, and healthdirect does not endorse any clinic, surgeon or treatment package.

Summaries and key points are condensed from the healthdirect page for readability. They are not a substitute for the original document, which was last reviewed in November 2025 and may have been updated since this library was compiled on 2026-10-20. Always read the current version on healthdirect.gov.au and discuss it with a qualified clinician.

Coverage

What the healthdirect information covers

The library is grouped into five areas, from what the procedure is through to recovery, risks and alternatives.

The procedure

What breast augmentation is, how implants are placed, anaesthetic, fat grafting and the TGA status of breast implant products in Australia.

Decisions and consent

Benefits people expect, the points healthdirect says to weigh up, and the questions to ask before surgery.

Preparation and cost

Getting fit, quitting smoking, GP referral, checking your surgeon on AHPRA and RACS, Medicare and the components of cost.

Recovery

Time off work, exercise limits, drains and dressings, and the symptoms that mean calling your surgeon or going to hospital.

Complications

Bleeding, infection, sensation changes, rupture, and the rarer serious risks including clots and implant-associated cancer.

Alternatives and support

Counselling, padded bras and non-implant surgery, plus Australian resources and the 24/7 healthdirect helpline.

All sections in this library

Grouped by topic. Each entry gives a short summary, the main extracted points and a direct link to that section of the healthdirect page.

What breast augmentation is

How healthdirect describes the procedure itself, why people choose it, and the points it says to weigh up before deciding.

What is breast augmentation?

Procedure page section · Reviewed November 2025

healthdirect defines breast augmentation as a form of cosmetic surgery that enlarges the breasts, also called breast enlargement or breast reconstruction. Implants sit either under the breast or under the chest muscle and consist of a silicone shell filled with saline (salty water) or silicone gel. Some surgeons also offer fat grafting, moving fat from another part of the body into the breast. The page notes that reconstruction after mastectomy is a separate topic, and that a Therapeutic Goods Administration (TGA) review led to some breast implants being withdrawn from the Australian market or given conditions on their use.

- Breast augmentation is cosmetic surgery to make the breasts larger; it is also referred to as breast enlargement.
- Implants are placed under the breast or beneath the chest muscle and are made of a silicone shell filled with saline or silicone gel.
- Some cosmetic surgeons offer fat grafting, transferring fat from areas such as the buttocks into the breasts.
- Reconstruction after a mastectomy uses implants or tissue for a different purpose; that is separate from cosmetic augmentation.
- After a TGA review, some breast implant products have been removed from the Australian market or had conditions placed on them.

Read the original on healthdirect.gov.au →

What are the benefits of breast augmentation?

Procedure page section · Reviewed November 2025

healthdirect states that people consider breast augmentation for many reasons, most commonly a desire to feel more physically attractive or more comfortable with themselves. The page frames the benefit in terms of how the person hopes to feel rather than promising any particular result.

- Reasons people give include wanting to feel more physically attractive or better about themselves.
- healthdirect does not guarantee an outcome; expectations should be discussed with a surgeon.

Read the original on healthdirect.gov.au →

What should I consider before choosing to have breast augmentation?

Procedure page section · Reviewed November 2025

healthdirect acknowledges social pressure around body shape and asks readers to weigh several points before deciding: implants are not lifelong and usually need replacing after 10 to 15 years; cancer has very rarely arisen in the fluid and scar tissue around an implant; some people are unhappy with how their breasts look afterwards; implants can make breast cancer harder to detect during screening; and breastfeeding may be affected.

- Implants do not last for life and usually need replacing after 10 to 15 years.
- There have been rare cases of cancer developing in the fluid and scar tissue around an implant.
- Some people find the result does not look the way they hoped.
- Implants can make it harder to detect breast cancer during screening checks.
- The ability to breastfeed may change after surgery.
- Body image and social pressure are worth examining before deciding, and counselling is an option.

Read the original on healthdirect.gov.au →

Current status of breast implant products in Australia

Regulator callout on the healthdirect page · Reviewed November 2025

The healthdirect page carries a callout about a Therapeutic Goods Administration (TGA) review after which some breast implants were removed from the Australian market or had conditions placed on them, and links to the TGA's current status page for the details.

- The TGA reviewed breast implant products and some were withdrawn from the Australian market or given conditions of use.
- The TGA website publishes the current status of breast implant products in Australia.
- The TGA also publishes information on breast implants and anaplastic large cell lymphoma (BIA-ALCL).

Read the original on healthdirect.gov.au →

The procedure and preparation

Anaesthetic and implant placement, how to prepare for surgery, choosing a surgeon through AHPRA and RACS, what Medicare covers, and the questions healthdirect suggests asking before surgery.

What does a breast augmentation procedure involve?

Procedure page section · Reviewed November 2025

The procedure is normally done under general anaesthetic so the patient sleeps throughout. The surgeon cuts alongside the breast and positions the implant either between the breast tissue and the muscle, or behind the large chest muscle under the breast; the surgeon decides which method suits the patient. Different surgical techniques are available, and the method chosen can affect recovery time.

- General anaesthetic is usually required, so you are asleep during the operation.
- The surgeon makes a cut alongside the breast.
- The implant is placed either between the breast tissue and the muscle, or behind the chest muscle.
- The surgeon chooses the method, and the technique used can affect how long recovery takes.

Read the original on healthdirect.gov.au →

How can I prepare for a breast augmentation?

Procedure page section · Reviewed November 2025

healthdirect advises gathering as much information as possible and preparing by getting physically fit, reaching a healthy weight, trying to stop smoking, asking about medication changes, disclosing health conditions and allergies, and confirming whether you will stay overnight. On choosing a surgeon, it says to pick someone you feel comfortable with who will be honest about benefits and risks, notes that a GP must refer cosmetic surgery patients to their surgeon, and points to the AHPRA register where you can confirm the surgeon is a Fellow of the Royal Australasian College of Surgeons (RACS) and check for serious disciplinary action. On cost, Medicare generally does not cover cosmetic surgery, private health insurance should be checked, and fees typically include the surgeon, anaesthetist, clinic or hospital, the implants, and medication, dressings and support garments. The page lists questions to ask about implant type, sensation, nipple position and appearance, bruising and pain, plus long-term questions about implant lifespan, breastfeeding, rupture, the likelihood of further operations and the cost of any repeat surgery.

- Prepare by getting physically fit, maintaining a healthy weight and trying to quit smoking.
- Ask whether any medications need to change, tell the surgeon about health conditions and allergies, and confirm whether you will stay overnight or go home.
- Choose a surgeon you feel comfortable with who will discuss benefits and risks honestly.
- A GP needs to make the referral to the surgeon you choose for cosmetic surgery.
- Check the AHPRA register to confirm your surgeon is a Fellow of the Royal Australasian College of Surgeons (RACS) and to see whether serious disciplinary action has been taken.
- Medicare generally does not cover cosmetic surgery; check your private health policy for any cover.
- Costs usually include surgeon and anaesthetist fees, clinic or hospital fees, the implants, and medication, dressings and support garments.
- Ask what implant type is recommended and why, whether sensation will change, how the nipples will be affected, and what bruising, swelling and pain to expect.
- Ask how long implants last, whether you will be able to breastfeed, how many of the surgeon's patients have had a ruptured or failed implant, how likely further operations are, and whether extra surgery would cost more.

Read the original on healthdirect.gov.au →

Recovery and warning signs

What to expect after surgery, the support typically needed while healing, and the symptoms that mean you should contact your surgeon or go to hospital.

How soon will I recover?

Procedure page section · Reviewed November 2025

Recovery depends on your health before surgery and your surgeon's advice; you may be asked to take a week off work and avoid moderate exercise for around five weeks. Tubes draining fluid, dressings and a compression bandage are common, along with pain and discomfort from swelling and bruising. Pain relief, extra support garments, time off work, help at home and a follow-up visit are often needed. healthdirect says to contact your surgeon or go to a hospital emergency department if you develop fever, severe pain, nausea or vomiting during recovery.

- Recovery varies with your pre-surgery health and your surgeon's advice.
- You may be asked to take about a week off work and avoid moderate exercise for around five weeks.
- Drain tubes, dressings and a compression bandage may be used, and pain, swelling and bruising are normal early on.
- You may need pain relief, support garments for a few weeks, time off work, help with daily tasks and a follow-up appointment.
- Fever, severe pain, nausea or vomiting during recovery means you should contact your surgeon or go to a hospital emergency department.

Read the original on healthdirect.gov.au →

When should I see my doctor?

Procedure page section · Reviewed November 2025

healthdirect lists the symptoms that mean contacting your surgeon straight away after surgery: a temperature above 38°C or chills; nausea, vomiting, shortness of breath or diarrhoea; heavy bleeding from the wound; bleeding or fluid leakage that starts more than a day after surgery; redness around the implant that is worsening or spreading; and increasing pain or tenderness. In an emergency, go to hospital or call triple zero (000).

- Contact your surgeon straight away for a temperature over 38°C or chills.
- Nausea, vomiting, shortness of breath or diarrhoea after surgery need urgent review.
- Heavy bleeding from the wound, or bleeding or fluid leakage starting more than one day after surgery, needs urgent review.

- Redness around the implant that worsens or spreads, and increasing pain or tenderness, should be reported.
- Fever, vomiting or heavy bleeding also warrants going to your nearest hospital.

Read the original on healthdirect.gov.au →

Complications and risks

The possible problems healthdirect lists after breast implant surgery, from bleeding and infection to rarer serious risks.

What complications can happen?

Procedure page section · Reviewed November 2025

Like any operation, breast implant surgery carries risks that should be discussed in detail with a surgeon. healthdirect lists bleeding or a haematoma, infection, changed sensation or numbness in the breast skin or nipple, swelling or a fluid collection, injury to surrounding tissue and implant rupture. Rarer serious risks include an allergic reaction to the anaesthetic, blood clots in the legs that can travel to the lungs, and breast implant-associated cancer — implants previously linked with cancer are no longer available. Severe chest pain or trouble breathing means calling triple zero (000) for an ambulance.

- Possible problems include bleeding or a haematoma, infection, altered sensation or numbness, swelling or fluid collections, injury to nearby tissue and implant rupture.
- Rarer serious risks include allergic reaction to the anaesthetic and blood clots in the legs that can travel to the lungs.
- Breast implant-associated cancer is listed as a rare serious risk; implants linked with cancer in the past are no longer available.
- Risks should be discussed in detail with your surgeon before you consent.
- Trouble breathing or severe chest pain requires calling triple zero (000) for an ambulance.

Read the original on healthdirect.gov.au →

Alternatives and Australian resources

Options other than implants, plus the Australian organisations, fact sheets and helplines healthdirect points readers to.

Are there alternatives to breast augmentation?

Procedure page section · Reviewed November 2025

healthdirect states that breast augmentation may not be right for everyone and lists counselling to help with accepting your body shape, wearing a padded bra, and other types of surgery that do not involve implants as alternatives.

- Counselling to help you accept your body shape is offered as an alternative.
- Wearing a padded bra is a non-surgical alternative.
- Other surgery that does not use implants may also be an option.

Read the original on healthdirect.gov.au →

Resources and support

Procedure page section · Reviewed November 2025

healthdirect recommends discussing the decision with your doctor first and links to the Australian Society of Plastic Surgeons, the Agency for Clinical Innovation breast implant safety patient information sheet, an Australian Government fact sheet on cosmetic surgery, and the TGA guide to the five questions to ask before getting a medical implant (available in community languages). It also gives the healthdirect helpline on 1800 022 222 (NURSE-ON-CALL in Victoria), staffed by registered nurses 24 hours a day, seven days a week.

- Discuss the decision with your doctor before going further.
- The Australian Society of Plastic Surgeons and the Agency for Clinical Innovation publish patient information on breast implants.
- The Australian Government has a cosmetic surgery fact sheet, and the TGA publishes a guide to questions to ask before a medical implant.
- The healthdirect helpline (1800 022 222, NURSE-ON-CALL in Victoria) is staffed by registered nurses 24/7.

Read the original on healthdirect.gov.au →

Extracted data

Key facts from the healthdirect page

The statements below summarise what healthdirect says about breast augmentation and related decisions. The right-hand column links to the source section.

Topic What healthdirect says Source What the procedure is Breast augmentation is a form of cosmetic surgery that enlarges the breasts; it is also called breast enlargement, and uses implants placed under or behind the breast. What is breast augmentation? Implant construction Breast implants are a silicone shell filled with either saline (salty water) or silicone gel, positioned under the breast or beneath the chest muscle. What is breast augmentation? Fat grafting Some cosmetic surgeons also offer fat grafting, in which fat is taken from another part of the body, such as the buttocks, and transferred into the breasts. What is breast augmentation? TGA market action Following a Therapeutic Goods Administration review, some breast implants have been removed from the Australian market or had conditions placed on them; current status is published on the TGA website. Current status of breast implant products in Australia Reasons people choose it People consider breast augmentation for many reasons, most often to feel more physically attractive or better about themselves. What are the benefits of breast augmentation? Implant longevity Breast implants do not last for life and usually need to be replaced after 10 to 15 years. What should I consider before choosing to have breast augmentation? Rare cancer risk There have been rare cases of cancer growing in the fluid and scar tissue around a breast implant. What should I consider before choosing to have breast augmentation? Screening Breast implants can make breast cancer harder to detect during screening checks, so continuing screening remains important. What should I consider before choosing to have breast augmentation? Breastfeeding The ability to breastfeed may change after breast augmentation surgery. What should I consider before choosing to have breast augmentation? Satisfaction Some people find that after surgery their breasts do not look the way they hoped, which is a reason to discuss expectations carefully before consenting. What should I consider before choosing to have breast augmentation? Anaesthetic Breast augmentation normally requires a general anaesthetic, so the patient is asleep during the operation. What does a breast augmentation procedure involve? Implant placement The implant is placed either between the breast tissue and the muscle or behind the large chest muscle under the breast, and the surgeon decides which method is right for you; the technique chosen can affect recovery time. What does a breast augmentation procedure involve? Preparation Before surgery, healthdirect advises getting physically fit, having a healthy body weight, trying to quit smoking, asking about medication changes, disclosing health conditions and allergies, and confirming whether you will stay overnight. How can I prepare for a breast augmentation? Choosing a surgeon healthdirect advises choosing a surgeon you feel comfortable with who will speak honestly about benefits and risks, and verifying credentials on the AHPRA register of medical practitioners. How can I prepare for a breast augmentation? GP referral Your general practitioner needs to make the referral to the surgeon you choose for cosmetic surgery, and your GP can help you decide. How can I prepare for a breast augmentation? Surgeon credentials On the AHPRA register you can check that your surgeon is a Fellow of the Royal Australasian College of Surgeons (RACS) and see whether serious disciplinary action has been taken against them. How can I prepare for a breast augmentation? Medicare and insurance Medicare generally does not cover the cost of cosmetic surgery; if you have private health insurance you should check your policy to see whether it covers the procedure. How can I prepare for a breast augmentation? What the cost includes The cost of a breast augmentation varies by patient and typically includes surgeon and anaesthetist fees, clinic or hospital fees, the implants, and medication, dressings and support garments, all of which should be discussed with your surgeon beforehand. How can I prepare for a breast augmentation? Questions to ask healthdirect suggests asking which implant type is recommended and why, whether sensation will change, how the position and appearance of the nipples will be affected, and how much bruising, swelling and pain to expect. How can I prepare for a breast augmentation? Long-term questions Long-term questions to

ask include how long the implants last, whether you will be able to breastfeed, how many patients the surgeon has seen with a ruptured or failed implant, how likely further operations are, and whether further surgery would cost extra. How can I prepare for a breast augmentation? Recovery time Recovery depends on your health before surgery and your surgeon's advice; you may be asked to take about a week off work and avoid moderate exercise for five weeks. How soon will I recover? After surgery After the operation you might have drain tubes, dressings, a compression bandage and pain and discomfort from swelling and bruising, and may need pain relief, support garments, help with daily activities and a follow-up appointment. How soon will I recover? Complications Possible complications include bleeding or a haematoma, infection, changed sensation or numbness in the breast skin or nipple, swelling or fluid collection, injury to surrounding tissue and implant rupture. What complications can happen? Serious risks Rarer serious risks include allergic reaction to the anaesthetic, blood clots in the legs that can travel to the lungs, and breast implant-associated cancer; implants previously linked with cancer are no longer available. What complications can happen? Emergency symptoms Trouble breathing or severe chest pain after surgery means calling triple zero (000) and asking for an ambulance. What complications can happen? When to call your surgeon Contact your surgeon straight away for a temperature over 38°C or chills, nausea, vomiting, shortness of breath or diarrhoea, heavy bleeding, bleeding or fluid leakage starting more than a day after surgery, worsening or spreading redness around the implant, or increasing pain or tenderness. When should I see my doctor? Recovery emergencies During recovery, fever, severe pain, nausea or vomiting mean you should contact your surgeon or go to a hospital emergency department. How soon will I recover? Alternatives Alternatives to breast augmentation listed by healthdirect are counselling to help you accept your body shape, wearing a padded bra, and other types of surgery that do not involve implants. Are there alternatives to breast augmentation? Australian resources healthdirect points to the Australian Society of Plastic Surgeons, the Agency for Clinical Innovation breast implant safety sheet, an Australian Government cosmetic surgery fact sheet and the TGA's guide to questions to ask before a medical implant. Resources and support Helpline The healthdirect helpline on 1800 022 222 (NURSE-ON-CALL in Victoria) connects you with a registered nurse 24 hours a day, seven days a week. Resources and support Clinical review The healthdirect breast augmentation page was last reviewed in November 2025 and its content is developed and reviewed under healthdirect's published content process. What is breast augmentation? Statements are paraphrased from healthdirect patient information (last reviewed November 2025). The page is reviewed periodically; verify against the linked healthdirect.gov.au section before relying on any point.

Jurisdiction and copyright

healthdirect pages describe practice, costs and regulation in Australia. Medicare, private insurance, GP referral rules, AHPRA registration and TGA device decisions are Australian arrangements; standards differ between countries and local rules still apply wherever a procedure is performed.

Summaries and facts on this page are paraphrased from healthdirect patient information published by the Australian Government. This library stores only summaries, extracted facts and links; refer to healthdirect’s content policy for the original terms.

For UK patient information, see the NHS information library . For US regulator guidance, see the FDA information library . For professional-society patient information on breast augmentation, see the ASPS information library . For published research, see the PMC research library .

Evidence base

Sources cited by the healthdirect page

healthdirect lists these publications and regulator pages behind its breast augmentation article.

[Image: Layered breast contour artwork on a deep burgundy background]

Breast Implants — Australian Journal of General Practice

Linked from the healthdirect breast augmentation page.

[Image: Abstract breast anatomy artwork with soft rose tissue forms]

Breastfeeding After Breast Augmentation Surgery: A Scoping Review

Linked from the healthdirect breast augmentation page.

[Image: Bold abstract breast contour lines on a burgundy background]

The Psychological Impact of Aesthetic Surgery: A Mini-Review

Linked from the healthdirect breast augmentation page.

[Image: Bold abstract breast contour lines on a burgundy background]

Advantages and Disadvantages of Breast Augmentation (Cureus)

Linked from the healthdirect breast augmentation page.

[Image: Bold abstract breast contour lines on a burgundy background]

Social Media Influence on Body Image and Cosmetic Surgery Considerations (Cureus)

Linked from the healthdirect breast augmentation page.

[Image: Abstract breast anatomy artwork with soft rose tissue forms]

Complications of reconstructive and aesthetic breast surgery (UpToDate)

Linked from the healthdirect breast augmentation page.

[Image: Breast-focused medical editorial artwork with bold gold and rose arcs]

Breast Implants: Common Questions and Answers (American Family Physician)

Linked from the healthdirect breast augmentation page.

[Image: Breast-focused medical editorial artwork with bold gold and rose arcs]

Breast implants and anaplastic large cell lymphoma (TGA)

Linked from the healthdirect breast augmentation page.

[Image: Breast-focused medical editorial artwork with bold gold and rose arcs]

Current status of breast implant products in Australia (TGA)

Linked from the healthdirect breast augmentation page.

[Image: Layered breast contour artwork on a deep burgundy background]

Five questions to ask your health professional before you get a medical implant (TGA)

Linked from the healthdirect breast augmentation page.

[Image: Abstract breast anatomy contours in rose against a dark burgundy ground]

Breast implant safety patient information sheet (Agency for Clinical Innovation)

Linked from the healthdirect breast augmentation page.

[Image: Layered breast contour artwork on a deep burgundy background]

Patient information sheet: cosmetic surgery (Australian Government)

Linked from the healthdirect breast augmentation page.

[Image: Breast-focused medical editorial artwork with bold gold and rose arcs]

Breast augmentation (Australian Society of Plastic Surgeons)

Linked from the healthdirect breast augmentation page.

[Image: Bold abstract breast contour lines on a burgundy background]

AHPRA register of medical practitioners

Linked from the healthdirect breast augmentation page.

[Image: Abstract breast-medical illustration with rose and gold contour lines]

FAQs — cosmetic surgery: requirement for GP referral (Medical Board of Australia)

Linked from the healthdirect breast augmentation page.

Related healthdirect pages

Other procedure pages on healthdirect that sit alongside breast augmentation:

- Breast reduction
- Breast uplift
- Breast reconstruction with an implant or tissue expander
- Breast reconstruction with abdominal tissue flap
- Breast reconstruction with latissimus dorsi flap
- Guide to cosmetic surgery
- Preparing for surgery
- Mammography

Download and reuse

The structured dataset behind this page is available as JSON . It contains the section list, categories, summaries, key points, extracted facts, cited references and related pages, with source URLs.

Your journey starts with information

Want to discuss what the healthdirect guidance means for you?

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Research Hub: Every Evidence Library on This Site

Page: /research/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Evidence

Research and Reference Libraries

This site is built from primary sources rather than opinion. Here is every library it draws on, what each one contains, and how to read it responsibly.

A claim is only as good as the source behind it. Rather than asking you to trust this website, the libraries below let you go straight to the original material — regulator guidance, public-health information, systematic reviews and the peer-reviewed record.

Primary evidence

Research collections

[Image: Abstract breast anatomy artwork with soft rose tissue forms]

PubMed

PubMed research

Curated, topic-grouped summaries of the peer-reviewed breast enlargement studies most relevant to patients — fat transfer, implants, external expansion, fillers and global trends — with a link to every record.

[Image: Breast-focused medical editorial artwork with bold gold and rose arcs]

PubMed Central

PMC research library

Every PubMed Central record for the exact phrase “breast enlargement”, searchable by title, abstract, author, journal and year. A complete capture, not a hand-picked list.

[Image: Breast-focused medical editorial artwork with bold gold and rose arcs]

Cochrane Library

Cochrane evidence library

Independent systematic reviews and registered protocols on breast implants, wound drains, implant types and reconstruction, with key findings and evidence gaps.

Regulator and public-health information

Official sources

[Image: Breast-focused medical editorial artwork with bold gold and rose arcs]

U.S. FDA

FDA information

US regulator pages on breast implants, dermal fillers and injectable-silicone warnings, summarised and linked.

[Image: Abstract breast anatomy contours in rose against a dark burgundy ground]

NHS (UK)

NHS information

UK public-health patient information on breast enlargement, costs, complications and choosing a practitioner.

[Image: Breast-focused medical editorial artwork with bold gold and rose arcs]

healthdirect (AU)

healthdirect information

Australian Government health advice on breast augmentation — preparation, costs, recovery, complications and alternatives.

[Image: Bold abstract breast contour lines on a burgundy background]

MedlinePlus (NLM)

MedlinePlus information

US National Library of Medicine consumer health information on breast augmentation surgery, risks, recovery and wound care.

Professional societies and academic sources

Clinical and society libraries

[Image: Abstract breast anatomy artwork with soft rose tissue forms]

BAPRAS (UK)

BAPRAS patient guide

UK professional-association guide to breast augmentation: consultation and cooling-off, implant techniques, complication rates, long-term consequences and the implant registry.

[Image: Breast-focused medical editorial artwork with bold gold and rose arcs]

BAAPS (UK)

BAAPS information

British Association of Aesthetic Plastic Surgeons patient information on breast augmentation and its risks.

[Image: Abstract breast anatomy contours in rose against a dark burgundy ground]

ASPS (US)

ASPS information

American Society of Plastic Surgeons patient information on breast augmentation: candidacy, cost, implant types, safety, recovery, results and choosing a surgeon.

[Image: Abstract breast anatomy artwork with soft rose tissue forms]

Cleveland Clinic

Cleveland Clinic information

Academic patient education on breast augmentation, implant types, fat transfer, recovery and complications.

[Image: Abstract breast anatomy contours in rose against a dark burgundy ground]

StatPearls

NCBI Bookshelf (StatPearls)

A peer-reviewed clinical review of breast augmentation — indications, contraindications, technique, complications and outcomes.

[Image: Abstract breast anatomy artwork with soft rose tissue forms]

Wikipedia

Wikipedia captures

Fixed-revision captures of the Wikipedia articles on breast enlargement and breast augmentation, with extracted facts and references, clearly labelled as encyclopedia content.

How to read research responsibly

An archive is not a recommendation service. A published paper can be small, observational, outdated or contradicted by later work. Systematic reviews and randomised trials sit at the top of the evidence ladder; single-centre case series and manufacturer surveys sit near the bottom. Before changing a treatment decision, read the full article, check its date and design, and discuss it with a qualified clinician.

Our own guides

Start with the summaries

[Image: Bold abstract breast contour lines on a burgundy background]

What works: evidence guide

A scorecard of more than 20 popular methods — what adds real volume, what only changes appearance, and what to avoid.

[Image: Breast anatomy-inspired abstract contours with luminous rose highlights]

References & sources

A single consolidated bibliography of every source this website indexes, grouped by publisher.

Your journey starts with information

Bring your findings to a qualified clinician

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

MedlinePlus Breast Enlargement Information | Breast Augmentation, Recovery & Risks | Breast Enlargement Guide

Page: /research/medlineplus-breast-enlargement/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Consumer health library

MedlinePlus Breast Enlargement Information

Breast enlargement information published by MedlinePlus, the U.S. National Library of Medicine — breast augmentation surgery, anaesthetic and incision options, risks, recovery and wound care, extracted and linked to the original medlineplus.gov pages.

This library collects MedlinePlus's public consumer-health information on breast enlargement: the breast augmentation operation itself, recovery and wound-care instructions, related breast procedures and the wider plastic and cosmetic surgery topic page. Every entry links to the original page on medlineplus.gov , so the wording, review dates and any subsequent updates can be checked at source.

What this library does and does not mean

MedlinePlus is a consumer-health information service of the U.S. National Library of Medicine. It does not provide medical advice, endorse any clinic, surgeon or treatment package, or approve breast enlargement as a procedure. Nothing here is a recommendation to have a treatment, and individual suitability can only be assessed in a consultation.

Summaries and key points are condensed from MedlinePlus pages for readability. They are not a substitute for the original documents, which may have been updated since this library was compiled on 2026-10-20. Always read the current version on medlineplus.gov and discuss it with a qualified clinician.

Coverage

What the MedlinePlus information covers

The library is grouped into five areas, from the operation itself to recovery, related procedures and visual resources.

The procedure

What breast augmentation is, implant fills and placement, anaesthetic choices, the four incision routes and what implant choice affects.

Risks and safety

General and breast-specific risks, capsular contracture, breast implant-associated lymphoma and the reported systemic symptoms called breast implant illness.

Recovery and wound care

Discharge instructions: activity and driving limits, dressings, drains, medicines and scar care, plus general care for an open surgical incision.

Before surgery

Screening mammograms, disclosing pregnancy and all medicines, managing chronic conditions, stopping

smoking and pausing blood thinners.

Related procedures

Breast lift and breast reduction articles, the illustrated image series and the breast lift anatomy video.

Topic hub

The Plastic and Cosmetic Surgery health topic page with statistics, clinical-trial searches, patient handouts and Spanish versions.

All MedlinePlus pages in this library

Grouped by topic. Each entry gives a short summary, the main extracted points and a direct link to the full MedlinePlus page.

Breast augmentation surgery

The MedlinePlus encyclopedia article on the operation itself: what it is, how implants are placed, the anaesthetic and incision options, why it is performed, and the full risk list.

Breast augmentation surgery

Medical Encyclopedia article · Page last reviewed 2025-02-17

The MedlinePlus overview of breast augmentation: enlarging or reshaping the breasts by placing an implant behind the breast tissue or under the chest muscle fascia. The article covers the two implant fills (sterile saline and silicone), the outpatient or hospital setting, general versus local anaesthetic, the four usual incision routes, how implant and technique choices affect pain, appearance, implant integrity and future mammograms, the reasons for surgery, the risks (including capsular contracture, implant rupture, breast implant-associated lymphoma and reported systemic symptoms), what to arrange before the operation, what happens afterwards, and the expected outcome.

- Augmentation is done by placing an implant behind the breast tissue or beneath the chest muscle, and implants are sacs filled with either sterile salt water or silicone.
- The operation is performed at an outpatient surgery clinic or in a hospital, most often under general anaesthetic, with local anaesthetic as an alternative.
- Four incision routes are described: the natural fold under the breast (the most common), the armpit using an endoscope, the edge of the areola, and, for saline implants only, a route near the navel through which the empty implant is tunnelled and then filled.
- An areola-edge incision may bring more difficulty with breastfeeding and more loss of nipple sensation, and may carry a higher risk of capsular contracture.
- Implant type, implant size and the chosen operation influence post-operative pain, the appearance achieved, the chance of later breakage or leakage, and the extra views future mammograms will need.
- Reasons for the procedure include increasing breast size, changing breast shape and correcting a congenital deformity; patients are told to expect improvement rather than perfection.
- Risks shared with any operation are listed as reactions to medicines, breathing problems, bleeding, blood clots and infection.
- Breast-specific risks include difficulty breastfeeding, numbness around the nipple or surrounding skin, small or thickened raised scars, uneven nipple position, differences in size or shape between the two breasts, implant breakage or leakage, visible rippling, infection requiring implant removal, the need for more breast surgery and puncture of the lung.
- A capsule of scar tissue normally forms around a new implant; when it thickens it can change the breast's shape, harden the breast tissue or cause pain.
- A rare immune-system cancer, breast implant-associated lymphoma, has been reported with some implants, presenting as a mass or swelling around the implant with breast pain and treated by removing the implant and

the tissue around it.

- Some people report systemic symptoms after implants — grouped under breast implant illness, which the page says is not yet established as a recognised illness — including fatigue, memory and concentration problems, joint and muscle pain, anxiety, hair loss, depression, rash, autoimmune disease, inflammation and weight change; some improve after removal and some do not.
- Before surgery patients should ask about a screening mammogram, disclose any pregnancy possibility and every medicine or supplement they take, review diabetes, heart disease and other conditions, stop smoking (which slows healing and raises clot risk), prepare the home and arrange transport and help for one to two days.
- During the week before surgery, blood thinners such as aspirin, ibuprofen, naproxen and vitamin E are commonly paused on the surgeon's advice, and fasting and morning-medicine instructions come from the surgical team.
- Afterwards the breasts are wrapped in bulky gauze or a surgical bra, drainage tubes are removed within about three days, and massaging from around day five may be suggested to reduce capsular hardening after checking with the provider.
- Outcomes are usually very good, scars are permanent and most visible during the first year before fading, a supportive bra may be worn for a few months, and later tests may be advised to confirm the implants are intact.

Read the original on medlineplus.gov →

Recovery, discharge and wound care

Patient instructions for the days and weeks after cosmetic breast surgery, plus general guidance for looking after a surgical incision that has been left open.

Cosmetic breast surgery - discharge

Patient instructions · Page last reviewed 2025-02-17

Home-care instructions after cosmetic breast surgery — augmentation, lift or reduction. It covers what happens in hospital, what to expect once home, activity and driving limits, dressing and wound care, pain medicines and general self-care, and the symptoms that should prompt a call to the surgical team.

- Surgery is done under general or local anaesthetic and takes an hour or more; patients wake with a gauze dressing or surgical bra and may have drainage tubes in place.
- Most people go home the same day, though some stay one to two nights in hospital.
- Pain, bruising and swelling are normal and settle over days to weeks, and reduced sensation in the breast skin or nipple may return gradually.
- Short walks are encouraged early to support circulation; returning to work or other activities commonly takes 7 to 14 days or longer.
- Heavy lifting, strenuous exercise and overstretching the arms are avoided for 3 to 6 weeks, and driving should wait at least two weeks and until drowsiness-inducing pain medicines have stopped.
- Drainage tubes are removed at a visit within a few days and stitches within two weeks; surgical glue dissolves on its own and does not need removing.
- Dressings stay in place as instructed, incisions are kept clean, dry and covered, and are checked daily for redness, pain or drainage.
- Once dressings are no longer needed, a soft wireless supportive bra is worn day and night for 2 to 4 weeks; bathing is limited to showering after drains come out, with baths, hot tubs and swimming deferred until stitches and drains are out and the surgeon agrees.
- Scars may take several months to more than a year to fade, and sunblock of SPF 30 or higher is advised whenever the scars are exposed.
- Smoking is not permitted because it slows healing and raises the risk of complications and infection; alcohol is avoided while taking narcotic pain medicines, and aspirin, ibuprofen and other medicines or supplements need the surgeon's approval.
- The surgeon should be contacted for increasing pain, redness, swelling, yellow or green drainage, bleeding or

bruising at the incision, medicine side effects, a temperature of 100°F (38°C) or higher, numbness, loss of motion, or sudden swelling of the breast.

Read the original on medlineplus.gov →

Surgical wound care - open

Patient instructions · Page last reviewed 2026-01-01

General MedlinePlus instructions for looking after a surgical incision that has broken open and is being left to heal rather than re-closed with stitches. It explains healing from the inside out, hand hygiene before dressing changes, removing and replacing dressings, cleaning and irrigating the wound, disposal of used supplies, and the signs that mean a call to the provider. The breast augmentation article links this page for wound care after breast surgery.

- A wound left open heals from the inside out, and the dressing absorbs drainage so the surface skin does not close before the tissue underneath has filled in.
- Hands should be washed before every dressing change — an alcohol cleanser, or soap and water for 15 to 30 seconds including under the nails, with jewellery removed.
- The skin around the wound is cleaned with saline or mild soapy water; alcohol, peroxide, iodine and antibacterial cleansers are avoided because they can damage wound tissue and slow healing.
- Where irrigation is advised, a syringe of saline or soapy water is held about 2.5 to 15 cm from the wound to wash away discharge, then the area is patted dry.
- No lotion, cream or herbal remedy goes on or near the wound without the provider's agreement; dressings are used once only and soiled material is bagged before disposal.
- Contact the provider for more redness, pain, swelling or bleeding, a wound that looks larger, deeper, dried out or dark, thicker or foul-smelling discharge, or a temperature of 100.5°F (38°C) or higher.

Read the original on medlineplus.gov →

Related breast procedures

Encyclopedia articles on other cosmetic breast operations that MedlinePlus indexes alongside augmentation.

Breast lift

Medical Encyclopedia article

MedlinePlus Medical Encyclopedia article on breast lift (mastopexy), listed by MedlinePlus among the cosmetic breast procedures alongside augmentation and reduction, and referenced from the cosmetic breast surgery discharge instructions.

Read the original on medlineplus.gov →

Breast reduction

Medical Encyclopedia article

MedlinePlus Medical Encyclopedia article on breast reduction (reduction mammoplasty), indexed on the Plastic and Cosmetic Surgery topic page and referenced from the cosmetic breast surgery discharge instructions as a related procedure.

Read the original on medlineplus.gov →

Videos and image series

Illustrated step-by-step series and the anatomy video that MedlinePlus attaches to its breast procedure articles.

Breast augmentation - series

Medical Encyclopedia image series

Illustrated image series walking through breast augmentation, one of three breast procedure series (with lift and reduction) published in the MedlinePlus encyclopedia and linked from the breast augmentation article page.

Read the original on medlineplus.gov →

Breast lift (mastopexy) - series

Medical Encyclopedia image series

Illustrated image series covering breast lift (mastopexy), including the incision patterns used, published in the MedlinePlus encyclopedia.

Read the original on medlineplus.gov →

Breast reduction (mammoplasty) - series

Medical Encyclopedia image series

Illustrated image series covering breast reduction (mammoplasty) and its indications, published in the MedlinePlus encyclopedia.

Read the original on medlineplus.gov →

Breast lift (anatomy video)

Anatomy video

MedlinePlus anatomy video explaining the breast lift procedure, embedded and recommended from the breast augmentation article page.

Read the original on medlineplus.gov →

Plastic and cosmetic surgery topic page

The MedlinePlus health topic hub that gathers the breast articles, image series, videos, statistics, clinical-trial searches and patient handouts.

Plastic and Cosmetic Surgery

Health topic page · Page last reviewed 2024-12-17

The MedlinePlus health topic hub for plastic and cosmetic surgery. It summarises the body areas people most often have reshaped — including increasing, reducing or reshaping sagging breasts — and links out to the encyclopedia articles and image series, the breast lift video, ASPS plastic surgery statistics, ClinicalTrials.gov searches for breast implants and plastic surgery, and patient handouts including the breast augmentation article in this library.

- Breast surgery appears among the most commonly requested cosmetic changes: enlarging, reducing or reshaping sagging breasts.
- The page indexes the breast augmentation, lift and reduction encyclopedia articles, their image series and the breast lift anatomy video.
- It links to FDA breast implant information, American Society of Plastic Surgeons statistics and ClinicalTrials.gov searches for breast implants and plastic surgery procedures.
- English and Spanish versions exist for the topic page and most of the linked resources.

Read the original on medlineplus.gov →

Extracted data

Key facts from the MedlinePlus pages

The statements below summarise what MedlinePlus says about breast enlargement and the decisions around it. The right-hand column links to the source page.

Topic What MedlinePlus says Source What breast augmentation is MedlinePlus describes breast augmentation as a procedure to enlarge the breasts or change their shape, carried out by placing an implant behind the breast tissue or under the muscle fascia or chest muscle. Breast augmentation surgery Implant fills An implant is a sac filled with either sterile salt water (saline) or a material called silicone. Breast augmentation surgery Setting and anaesthetic The operation is done at an outpatient surgery clinic or in a hospital, with most women receiving general anaesthesia while local anaesthesia — staying awake with the breast area numbed — is an alternative. Breast augmentation surgery Incision routes Four approaches are described: a cut in the natural fold under the breast (the most common technique), a cut under the arm often using an endoscope (leaving no scar on the breast but possibly one under the arm), a cut around the areola edge, and, for saline implants, a cut near the navel through which the implant is moved up and filled. Breast augmentation surgery Areola-edge trade-offs The periareolar route may bring more problems with breastfeeding and loss of sensation around the nipple, and may carry a higher risk of capsular contracture. Breast augmentation surgery What changes the result The type of implant, its size and the type of surgery affect post-operative pain, the appearance of the breast, the risk of the implant breaking or leaking later, and the additional views future mammograms will require. Breast augmentation surgery Why it is performed The procedure is carried out to increase breast size, to change breast shape, or to correct a congenital deformity, with patients advised to discuss realistic expectations — improvement rather than perfection. Breast augmentation surgery General surgical risks Risks applying to anaesthesia and surgery in general include reactions to medicines, breathing problems, bleeding, blood clots and infection. Breast augmentation surgery Breast surgery risks Listed breast risks include difficulty breastfeeding, loss of feeling in the nipple area or surrounding skin, small or thickened raised scars, uneven nipple position, differences in size or shape between the breasts, breaking or leakage of the implant, visible rippling, removal of the implant because of infection, the need for further breast surgery and puncture of the lung. Breast augmentation surgery Capsular contracture A capsule of scar tissue normally forms around a new implant to hold it in place; if this capsule thickens and grows it can change the breast's shape, harden breast tissue or cause pain. Breast augmentation surgery Breast implant-associated lymphoma A rare lymphoma of the immune system has been reported with some implant types, presenting as a mass or swelling around the implant together with breast pain, and treated by removing the implant and the tissue around it; research into other cancer links is ongoing. Breast augmentation surgery Breast implant illness Some people report systemic symptoms after receiving implants — fatigue, memory and concentration problems, joint and muscle pain, anxiety, hair loss, depression, rash, autoimmune disease, inflammation and weight gain or loss — described as breast implant illness, which the page says does not yet have enough evidence to be a recognised illness; some report improvement after implant removal and others do not. Breast augmentation surgery Overall safety statement Despite these rare risks, MedlinePlus states that breast implants are considered safe. Breast augmentation surgery Screening before surgery Patients are advised to ask whether a screening mammogram is needed given their age and breast cancer risk, arranged early enough that any further imaging or biopsy cannot delay the planned operation. Breast augmentation surgery What to tell the surgeon The surgeon or nurse should be told about any pregnancy or possibility of pregnancy and about every medicine, drug, supplement or herb being taken, including items bought without a prescription. Breast augmentation surgery Medical conditions and smoking Diabetes, heart disease and other conditions may require review by the provider who treats them, and smoking should be cut down or stopped because it slows healing and raises the risk of blood clots — surgery may be postponed while smoking continues. Breast augmentation surgery Blood thinners before surgery During the week before surgery, medicines that stop blood clotting — including over-the-counter aspirin, ibuprofen, naproxen and vitamin E as well as many prescription medicines — are commonly stopped temporarily on the surgeon's advice, with instructions on which medicines to still take on the day of surgery. Breast augmentation surgery Day-of-surgery instructions On the day of surgery patients follow the rules given about when to stop eating and drinking, take only the prescribed medicines with a small sip of water, and arrive at the hospital at the requested time. Breast augmentation surgery Immediately after surgery Most patients go home once the anaesthetic has worn off and they can walk, drink water and use the bathroom safely; the breasts are wrapped in a bulky gauze dressing or a surgical bra, and drainage tubes, if placed, are removed within 3 days. Breast augmentation surgery Massage after surgery Massaging the breasts from about 5 days after surgery may be recommended to reduce hardening of the capsule around the implant, but should only begin after checking with the provider. Breast augmentation surgery Outcome and follow-up Outcomes from breast surgery are usually very good, pain and skin symptoms from the surgery generally settle, a special supportive bra may be worn for a few months, scars are permanent but typically fade after being most visible in the first year, and future tests may be recommended to confirm the implants have not ruptured. Breast augmentation surgery Activity after discharge Home instructions after cosmetic breast surgery allow short walks

early, put return to work or other activity at 7 to 14 days or longer, and prohibit heavy lifting, strenuous exercise and overstretching the arms for 3 to 6 weeks, with driving delayed at least 2 weeks. Cosmetic breast surgery - discharge Signs to call the surgeon The surgical team should be contacted for increasing pain, redness, swelling, yellow or green drainage, bleeding or bruising at the incision, medicine side effects, a fever of 100°F (38°C) or higher, numbness or loss of motion, or sudden swelling of the breast. Cosmetic breast surgery - discharge Scar care after discharge Incision scars may take several months to more than a year to fade, and protecting them with sunblock of SPF 30 or higher is advised whenever they are exposed to the sun. Cosmetic breast surgery - discharge Healing an open incision Where an incision has broken open and is not re-closed, healing proceeds from the inside out and a dressing absorbs drainage so the surface does not close over before the wound underneath has filled in. Surgical wound care - open Cleansers to avoid Saline or mild soapy water is used to clean around the wound, while skin cleansers, alcohol, peroxide, iodine and antibacterial soaps are avoided because they can damage wound tissue and slow healing. Surgical wound care - open Wound infection warnings A provider should be contacted if redness, pain, swelling or bleeding increases, the wound looks larger, deeper, dried out or dark, discharge becomes thicker or smells of pus, or temperature reaches 100.5°F (38°C). Surgical wound care - open Scope of the topic page The MedlinePlus Plastic and Cosmetic Surgery topic page lists increasing, reducing and reshaping sagging breasts among the most common cosmetic requests and indexes the breast augmentation, lift and reduction articles, image series and video. Plastic and Cosmetic Surgery
Statements are paraphrased from MedlinePlus consumer health information. Pages carry their own review dates and may be updated; verify against the linked medlineplus.gov page before relying on any point.

Jurisdiction and copyright

MedlinePlus pages describe care in the United States; costs, professional registration, screening programmes and clinical standards differ between countries, and local standards still apply where a procedure is performed.

This library stores only original summaries, extracted facts and links to publicly available MedlinePlus pages. Encyclopedia and patient-instruction content on medlineplus.gov is produced by A.D.A.M. for the U.S. National Library of Medicine and remains subject to the MedlinePlus and A.D.A.M. terms of use.

For regulator guidance, see the FDA information library and the NHS information library . For UK surgical-association patient guidance, see the BAAPS information library . For Australian Government health advice, see the healthdirect information library . For academic patient education, see the Cleveland Clinic information library . For published research, see the PMC research library .

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The structured dataset behind this page is available as JSON . It contains the page list, categories, summaries, key points and extracted facts, with source URLs.

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NHS Breast Enlargement Information | Breast Implants, Costs & Safety || Quantum Health

Page: /research/nhs-breast-enlargement/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Regulator library

NHS Breast Enlargement Information

Breast enlargement information published by the National Health Service — the operation, costs, implant choices, recovery, complications, BIA-ALCL, PIP implants and choosing a surgeon, extracted and linked to the original nhs.uk pages.

This library collects the NHS's public patient information on breast enlargement and the decisions around it. Every entry links to the original page on nhs.uk , so the wording, review dates and any subsequent updates can be checked at source.

What this library does and does not mean

The NHS is a national health service, not a regulator of cosmetic procedures. Its pages describe how breast enlargement is provided and funded in the UK, what the operation involves and what can go wrong. Nothing here is a recommendation to have a procedure, and the NHS does not endorse any clinic, surgeon or treatment package.

Summaries and key points are condensed from NHS pages for readability. They are not a substitute for the original documents, which may have been updated since this library was compiled on 2026-10-20. Always read the current version on nhs.uk and discuss it with a qualified clinician.

Coverage

What the NHS information covers

The library is grouped into five areas, from the procedure itself to decisions, practitioner checks, funding and surgery abroad.

The procedure

What breast enlargement involves, anaesthetic, operation time, implant types and placement, and how long implants last.

Recovery and risks

Time off work, driving and exercise limits, and the complications the NHS lists, from capsular contracture to rupture.

Safety concerns

BIA-ALCL, breast implant illness, PIP implants, the implant registry and reporting problems through the Yellow Card Scheme.

Decisions and consent

Questions to ask yourself, cooling-off periods, avoiding pressure, and what to ask a surgeon at consultation.

Funding and screening

When the NHS may fund breast enlargement, UK costs, and breast screening and mammograms after implants.

Practitioner checks

CQC clinic registration, the GMC medical register, RCS certification, and BAAPS, BAPRAS, JCCP and Save Face registers.

All NHS pages in this library

Grouped by topic. Each entry gives a short summary, the main extracted points and a direct link to the full NHS page.

Breast enlargement (implants)

The NHS page on breast enlargement: what the operation involves, costs, implant choices, longevity, recovery, complications, BIA-ALCL, PIP implants, the implant registry and breast screening.

Breast enlargement (implants)

Patient information page · Page last reviewed 2023-09-21

The NHS overview of breast enlargement (also called a boob job or breast augmentation). Implants are inserted to increase size, change shape or make breasts more even. The page covers funding, UK costs, choosing a surgeon and implants, how long implants last, the operation itself, recovery, complications, BIA-ALCL, breast implant illness, PIP implants, the implant registry, breast screening and how to report problems.

- Breast enlargement is major surgery; results are not guaranteed and there are risks to weigh up.
- It is usually paid for privately. The NHS may fund it in some circumstances, for example very uneven breasts or no breasts, depending on local rules.
- In the UK, breast implant surgery costs around £3,500 to £8,000, which does not usually include consultations or follow-up care.
- Two implant types are offered: silicone (most common in the UK) and saline. Placement is either behind the breast tissue or behind the chest muscle.
- Implants are not lifetime devices; some women need further surgery after about 10 years.
- The operation is done under general anaesthetic and takes between 60 and 90 minutes; most people go home the same day or stay overnight.
- Recovery takes a few weeks: one or two weeks off work, no driving for at least a week, and no heavy lifting or strenuous exercise for at least a month.
- Reported problems include capsular contracture, rupture, rippling, rotation, nipple sensation changes and reduced breast milk while breastfeeding.
- BIA-ALCL is an uncommon immune-system cell cancer associated with implants; breast implant illness is a term for symptoms some women report after surgery.
- PIP implants were banned in the UK in 2010; a ruptured implant should be removed as soon as possible.
- The Breast and Cosmetic Implant Registry (BCIR), set up in 2016, allows implants to be traced if a safety concern arises.
- Breast screening still matters after implants; mammograms are safe and can be performed differently so as much breast tissue as possible is seen.
- Check any hospital or clinic in England is registered with the CQC, and that the surgeon is on the GMC specialist register with a licence to practise; always meet the surgeon before the procedure.
- The MHRA publishes a Cosmetic Breast Augmentation Patient Risk Awareness Tool to help discuss implant risks and keep a record of treatment.

Read the original on nhs.uk →

Deciding whether to go ahead

NHS advice on working out whether a cosmetic procedure is right for you, and the steps to take before

committing to any procedure.

Is a cosmetic procedure right for me?

Advice page · Page last reviewed 2023-05-09

NHS advice on working out whether a cosmetic procedure is a sound decision: questions to ask yourself, how to research, speaking to an expert, taking time, avoiding pressure and considering mental health.

- Ask what you want to change and why, how long you have thought about it, what triggered it now, and whether there are other ways to achieve the same change.
- Ask whether you are expecting the procedure to change your life as well as your appearance, and whether you want it for yourself or to please someone else.
- Consider all costs, including aftercare, correcting problems and future procedures needed to maintain the look.
- You should be given a cooling-off period after your consultation to decide.
- Avoid time-limited offers, extra treatments you had not considered, group treatment events involving alcohol, and mobile services operating in private homes or hotels.
- Tell the person treating you for a mental health condition, and your cosmetic practitioner, about any mental health problems past or present.
- Free NHS talking therapies are available for anxiety and depression if appearance concerns are affecting your wellbeing.

Read the original on nhs.uk →

Before you have a cosmetic procedure

Advice page · Page last reviewed 2023-05-22

NHS checklist for people considering a cosmetic procedure, surgical (such as breast implants) or non-surgical (such as dermal fillers or Botox): be clear about why, speak to an expert, research, understand costs, choose a qualified practitioner, ask questions, be cautious with social media and take time.

- All cosmetic procedures carry some risks, and complications can be serious if they are not done correctly.
- A GP or mental health professional can be a good first conversation, alongside a consultation with the practitioner.
- Understand the full costs including aftercare and any future procedures needed to maintain the result.
- Choose a practitioner who is trained, experienced and has the right insurance to correct anything that goes wrong.
- Social media influencers are usually paid to advertise services, so research anything found there with extra care.
- You should be offered a cooling-off period after any consultation, and you can walk away at any point.

Read the original on nhs.uk →

Choosing who will do the procedure

How to check a surgeon or practitioner is properly qualified, registered and insured, and what to ask at a consultation.

Choosing who will do your cosmetic procedure

Advice page · Page last reviewed 2023-06-23

NHS guidance on selecting a practitioner with the right training, skills and insurance: what to ask at a consultation, what the practitioner should explain, checks for cosmetic surgery in England (CQC clinic registration, GMC medical register, RCS certification, BAAPS/BAPRAS membership) and registers for non-surgical practitioners.

- Always book a consultation with the person who will do the procedure before deciding.

- Ask how many procedures they have done, their qualifications, common complications, aftercare, what to do if something goes wrong, and the full cost including any further treatment.
- The practitioner should explain how the procedure is done, how long it takes, whether anaesthetic is needed, expected pain, recovery time, risks, how long results last and what to expect afterwards.
- Do not pay for a procedure until you have had a consultation.
- In England, all independent hospitals and clinics providing cosmetic surgery must be registered with the Care Quality Commission (CQC), which publishes quality ratings.
- Only registered doctors can perform cosmetic surgery in England; check the GMC medical register, and note the Royal College of Surgeons certifies cosmetic surgeons against a national standard.
- For non-surgical procedures, look for practitioners on PSA-accredited voluntary registers such as JCCP or Save Face.
- Avoid group treatments, treatment vouchers from discount sites, mobile services in homes or hotels, and practitioners who only advertise on social media.
- Dermal filler complications can be serious, including infection, nerve damage and blindness, so avoid practitioners with only short training courses.

Read the original on nhs.uk →

Cosmetic surgery on the NHS

When the NHS may fund cosmetic procedures such as breast enlargement, and how that differs from reconstructive surgery.

Is cosmetic surgery available on the NHS?

Advice page · Page last reviewed 2023-04-17

Cosmetic surgery is not routinely provided on the NHS, though it may occasionally be funded for psychological or other health reasons. Breast enlargement may be funded for very uneven breasts or no breasts causing significant psychological distress. Reconstructive plastic surgery, such as breast rebuilding after mastectomy or cleft lip and palate repair, is different and often available.

- Most people who want cosmetic surgery pay for it privately.
- NHS breast enlargement funding depends on local commissioning rules; your GP can explain the rules in your area.
- Other examples where the NHS may pay include nose reshaping for breathing problems, ear correction, and breast reduction causing backache.
- Male breast reduction may be funded for long-standing gynaecomastia that has not responded to other treatments and causes considerable pain or distress.
- Reconstructive surgery aims to repair or reconstruct missing or damaged tissue after illness, accident or birth defect, and is often available on the NHS.

Read the original on nhs.uk →

Cosmetic surgery abroad

NHS advice on weighing savings against risks when considering cosmetic surgery in another country, including aftercare and flying after surgery.

Cosmetic surgery abroad

Advice page · Page last reviewed 2022-12-14

NHS advice on cosmetic surgery abroad: lower prices must be weighed against risks, follow-up care may be missing or of a different standard, holiday packages need caution, and aftercare, insurance and flying plans should be sorted out in advance.

- Overseas clinics may not provide follow-up treatment, or not to the same standard as the UK, and may have no UK professional you can see if problems arise.
- Be cautious of websites selling cosmetic surgery as part of a holiday: meet the surgeon, and do not pay for a hospital you have never seen or a surgeon you have not met.
- Recovering straight after a holiday is unrealistic; rest is required, with no alcohol, sunbathing, sightseeing or swimming.
- Find out what happens if there are complications, where follow-ups will be, how easily you could travel back for more surgery, and how much it would cost.
- Standards and qualifications abroad may differ, so find out how surgeons and clinics are regulated in the chosen country.
- You should have two consultations with the treating surgeon before surgery, giving a cooling-off period; they should explain the procedure, its limitations, alternatives and risks, and show before-and-after pictures.
- Travel insurance normally does not cover things going wrong with surgery; check clinic insurance, travel cover and how you would get home in an emergency.
- Do not fly for 5 to 7 days after procedures such as breast surgery, as air travel plus major surgery increases the risk of a life-threatening blood clot.

Read the original on nhs.uk →

Extracted data

Key facts from the NHS pages

The statements below summarise what the NHS says about breast enlargement and related decisions. The right-hand column links to the source page.

Topic What the NHS says Source Funding Breast enlargement is usually paid for privately. The NHS may fund it in some circumstances, for example very uneven breasts or no breasts, and it often depends on the rules in your local area; your GP can explain them. Breast enlargement (implants) NHS funding exceptions Cosmetic surgery is not routinely provided on the NHS, but may be provided for psychological or other health reasons, such as breast enlargement causing significant psychological distress. Is cosmetic surgery available on the NHS? Cost in the UK Breast implant surgery costs around £3,500 to £8,000 in the UK. This does not usually include consultations or follow-up care, and future follow-up surgery is also payable. Breast enlargement (implants) What the operation does Implants are inserted to increase breast size, change their shape or make breasts more even. The procedure is often known as a boob job or breast augmentation. Breast enlargement (implants) Implant types There are two types of implant: silicone and saline. Silicone is the most common in the UK — less likely to wrinkle and feels more natural, but can spread into the breast and cause lumps. Saline is more likely to fold, rupture or deflate over time, but leaked saline is safely absorbed by the body. Breast enlargement (implants) Implant placement Implants are placed either behind the breast tissue or behind the chest muscle, and size, shape and placement are discussed with the surgeon before the operation. Breast enlargement (implants) Implant longevity Breast implants do not last a lifetime and will probably need replacing at some point. Some women need further surgery after about 10 years because of implant problems or because the breasts have changed around the implants. Breast enlargement (implants) The operation Breast implant surgery is usually done under general anaesthetic. An incision is made next to or below the breast, the implant is positioned, and the incision is stitched and dressed. The operation takes between 60 and 90 minutes, with same-day discharge or an overnight stay. Breast enlargement (implants) Recovery Take one or two weeks off work, do not drive for at least one week, and avoid heavy lifting or strenuous exercise for at least a month. Stitches come out after one to two weeks, most normal activity resumes after about six weeks, and breasts settle over a few months. Some surgeons recommend a sports bra 24 hours a day for up to three months. Breast enlargement (implants) Sunbathing and flying It is safe to sunbathe and fly if you have breast implants, once recovered. Separately, the NHS advises not flying for 5 to 7 days after breast surgery because of blood clot risk. Breast enlargement (implants) Common implant problems Breast implants can cause thick scarring, capsular contracture (the breast feeling hard as scar tissue shrinks around the implant), rupture, creases or folds, rotation, rippling, nipple nerve changes that may be temporary or permanent, and slightly less breast milk while breastfeeding. Breast enlargement (implants) General surgical risks As with any operation there is a small risk of bleeding and clots (blood clots can be life threatening), infection treated with antibiotics, and allergic

reaction to medicines or products used during surgery such as antibiotics or latex. Breast enlargement (implants) Rupture detection A ruptured implant may cause small tender lumps (siliconomas) visible only on breast scans, and the implant will need to be removed. Ultrasound or MRI scans can check whether implants have ruptured. Breast enlargement (implants) BIA-ALCL There is an association between breast implants and an uncommon immune system cell cancer called breast implant associated anaplastic large cell lymphoma (BIA-ALCL), which develops in the scar tissue around implants in a very small number of people. Breast enlargement (implants) Breast implant illness Some women report symptoms after breast implant surgery such as tiredness, anxiety and joint pain, sometimes referred to as breast implant illness. Breast enlargement (implants) PIP implants PIP (Poly Implant Prostheses) implants were banned in the UK in 2010 after they were found to contain unapproved silicone gel and to be more likely to split. Anyone with PIP implants should get medical advice, and a ruptured implant should be removed as soon as possible. Breast enlargement (implants) Implant registry The Breast and Cosmetic Implant Registry (BCIR) was set up in 2016 to record the details of anyone who has breast implants for any reason, so they can be traced if there is a safety concern about a specific implant type. Breast enlargement (implants) Screening after implants You can still get breast cancer after implants and should continue regular breast screening. Mammograms are safe and do not rupture implants; because X-rays cannot pass through implants, the mammogram may be done differently so as much breast tissue as possible can be seen. Breast enlargement (implants) Reporting problems Problems with breast implants can be reported through the government's Yellow Card Scheme. Concerns about care can go to the Care Quality Commission, and concerns about a doctor to the General Medical Council. Breast enlargement (implants) Choosing a clinic in England All independent clinics and hospitals providing cosmetic surgery in England must be registered with the Care Quality Commission (CQC), which publishes quality ratings online. Choosing who will do your cosmetic procedure Choosing a surgeon Check the surgeon is registered with the General Medical Council (GMC), listed on the specialist register and holding a licence to practise. Many are also members of BAPRAS or BAAPS, whose registers can be checked, and the Royal College of Surgeons certifies cosmetic surgeons against an agreed national standard. Choosing who will do your cosmetic procedure Questions for your surgeon The NHS suggests asking about qualifications and experience, professional association membership, how many breast enlargements they have performed, how many operations involved complications, the implant type, manufacturer and why it is used, the surgical technique and placement, expected results, follow-up if things go wrong, patient satisfaction rates and alternative options. Breast enlargement (implants) Risk awareness tool The Medicines and Healthcare products Regulatory Agency (MHRA) publishes a Cosmetic Breast Augmentation Patient Risk Awareness Tool (PDF) that patients can use to discuss potential implant risks with their surgeon and keep a record of their treatment. Breast enlargement (implants) Further information The NHS points to patient information from the British Association of Aesthetic Plastic Surgeons (BAAPS), the British Association of Plastic, Reconstructive and Aesthetic Surgeons (BAPRAS) and the Royal College of Surgeons for more on breast augmentation and cosmetic surgery. Breast enlargement (implants) Time to decide You should be given a cooling-off period after your consultation. The NHS warns against time-limited offers, extra treatments pushed at the clinic, group events involving alcohol, and mobile services operating in private homes or hotels. Is a cosmetic procedure right for me? Payment Do not pay for a procedure until you have had a consultation to make sure it is right for you. Choosing who will do your cosmetic procedure Social media advertising Be cautious about procedures or clinics found on social media; influencers are usually paid to advertise, and practitioners who only advertise on social media should be avoided. Before you have a cosmetic procedure Surgery abroad Cosmetic surgery abroad can cost less, but follow-up care may be missing or of a different standard, travel insurance normally does not cover surgical complications, and you should have two consultations with the surgeon before surgery. Do not fly for 5 to 7 days after breast surgery. Cosmetic surgery abroad

Statements are paraphrased from NHS patient information. Pages are reviewed periodically; verify against the linked nhs.uk page before relying on any point.

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For regulator guidance, see the FDA information library . For UK surgical-association patient guidance, see the BAAPS information library . For Australian Government health advice, see the healthdirect information library .

For academic patient education on augmentation, implants and fat transfer, see the Cleveland Clinic information library . For professional-society patient information on breast augmentation, see the ASPS information library . For U.S. consumer health information on the operation and recovery, see the MedlinePlus information library . For published research, see the PMC research library , or read our evidence guide . For professional-association patient guidance, see the BAPRAS patient guide .

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Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

NCBI Bookshelf Breast Augmentation Review (StatPearls) | Indications, Technique & Complications

Page: /research/statpearls-breast-augmentation/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Clinical review library

NCBI Bookshelf Breast Augmentation Review

The StatPearls clinical review of breast augmentation from NCBI Bookshelf — indications, contraindications, technique, complications and outcomes, extracted section by section and linked to the original.

This library collects the clinical review Breast Augmentation published on NCBI Bookshelf as part of StatPearls — peer-reviewed point-of-care reviews produced for the US National Library of Medicine. Every entry links back to the matching section of the original review, so wording, dates and any later updates can be checked at source.

About this source

Breast Augmentation , Dean Fardo; Miguel B. Sequeira Campos; Jay M. Pensler · King's College London; Northwestern University Feinberg School of Medicine. Bookshelf ID NBK482206 · PMID 29489168 · last updated 2023-07-18. Fardo D, Sequeira Campos MB, Pensler JM. Breast Augmentation. [Updated 2023 Jul 18]. In: StatPearls [Internet]. Treasure Island (FL): StatPearls Publishing; 2023 Jan-.

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Coverage

What the review covers

The review is grouped here into four themes, matching the sections of the original article.

Background

What augmentation is, the FDA history of silicone implants, monitoring advice, and the anatomy, blood supply and nerve supply of the breast.

Who it is for

Indications from reconstruction to developmental hypomastia and elective self-image, and the medical and psychological contraindications.

The operation

Equipment, the theatre team, saline and silicone implants, smooth and textured surfaces, placement, incisions, anaesthesia, duration and recovery.

Risks and results

Early and late complications, capsular contracture grading and microbiology, implant longevity, ALCL, reported volumes and satisfaction.

All sections of the review

Grouped by theme. Each entry gives a short summary, the main extracted points and a direct link to that section of the NCBI Bookshelf article.

Background and anatomy

What breast augmentation is, why it is performed, how the FDA has treated silicone implants over time, and the anatomy the operation works on. (3)

Continuing Education Activity

StatPearls review section · Last updated 2023-07-18

The review's opening summary: breast augmentation increases breast size, most often with an implant and less commonly with fat transfer, and is among the most commonly performed plastic surgery procedures in the United States. It frames the activity around indications, contraindications, complications and the role of the interprofessional team before and after surgery.

- Objectives are to identify the different techniques, describe the indications, review the complications and improve care coordination among the team.
- The FDA restricted silicone-filled implants for cosmetic enhancement from 1992; reconstructive patients continued to receive them after mastectomy.
- The activity is written for the interprofessional team involved in pre- and postoperative care.

Read this section on ncbi.nlm.nih.gov →

Introduction

StatPearls review section · Last updated 2023-07-18

Breast augmentation is a surgical procedure in which breast size is enhanced, either by placing an implant or, less commonly, through fat transfer. The section traces FDA policy on silicone implants from the 1992 restriction to the 2006 reversal, describes the shift towards cohesive gel and form-stable implants, and sets out monitoring advice and the fact that implants are not permanent devices.

- Extensive research failed to confirm the disease fears that led to the 1992 silicone restriction, and the FDA allowed general use of silicone-filled implants again in 2006.
- By 2017 the majority of patients undergoing augmentation received cohesive gel silicone implants, with makers introducing different levels of gel firmness.
- The FDA recommends MRI screening every two years in patients with silicone implants to detect subclinical leaks, but adherence is incomplete and MRI readings can be false positive.
- Higher gel cohesion may lower rupture rates, but the review states that eventually all implants will rupture or break.

Read this section on ncbi.nlm.nih.gov →

Anatomy and Physiology

StatPearls review section · Last updated 2023-07-18

The anatomy relevant to augmentation: the breast sits on the anterior chest wall below the clavicle, bounded medially by the sternum, laterally by the latissimus dorsi and inferiorly by the inframammary fold, with the pectoralis muscles forming its posterior boundary.

- The inframammary fold is a dermal structure formed by fusion of the superficial and mammary fascia.
- Blood supply comes from the subdermal plexus with major contributions from the internal mammary artery, external mammary artery and intercostal perforators.
- Nerve supply is mainly through the intercostal plexus, with most contribution from the third to fifth intercostal nerves.

Read this section on ncbi.nlm.nih.gov →

Indications and contraindications

Who the review says the procedure is for, and the conditions that should stop a surgeon from operating. (2)

Indications

StatPearls review section · Last updated 2023-07-18

Implants are placed for reconstruction after mastectomy, for developmental disorders such as hypomastia, and for elective reasons related to self-image. Hypomastia may follow normal development or involution, especially after pregnancy, and people of all personality types may seek the procedure.

- Reported motivations include reconstruction, developmental under-development of the breast and elective enhancement of self-image.
- The review notes that when surveyed after surgery, the vast majority of women would undergo the operation again.

Read this section on ncbi.nlm.nih.gov →

Contraindications

StatPearls review section · Last updated 2023-07-18

Most patients have no contraindications, but the review lists the conditions that should prevent surgery and the patient factors that should pause it.

- Absolute reasons not to operate include active infection of the breast, active cancer and pregnancy.
- Relative reasons include a history of autoimmune disease, current radiation treatment, unstable medical conditions and known sensitivity to silicone.
- Psychological instability or unrealistic expectations are also given as reasons not to proceed.

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The operation

Equipment, theatre team, preparation, implant choices, placement, incisions, anaesthesia, duration and expected recovery. (4)

Equipment

StatPearls review section · Last updated 2023-07-18

Routine operating-room equipment is usually sufficient. Extras depend on the approach: a lighted retractor or headlight for visualising the pocket, an endoscope for umbilical or axillary approaches, and sometimes an implant sleeve used as part of a no-touch technique.

- A no-touch technique limits handling of the implant to reduce implant trauma and bacterial contamination.
- Endoscopic equipment may be required when the umbilical or axillary route is used.

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Personnel

StatPearls review section · Last updated 2023-07-18

The operating team normally consists of the surgeon, a scrubbed assistant, an anaesthesiologist and a circulating nurse working together in the operating room.

- Standard theatre roles apply: surgeon, scrubbed assistant, anaesthesiologist and circulating nurse.
- The team structure is described as the same as for other routine surgical procedures.

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Preparation

StatPearls review section · Last updated 2023-07-18

Patients are prepared for surgery as for any other surgical procedure, following the usual preoperative pathway.

- Preparation follows the routine preoperative work-up used for any operation, including assessment and consent.
- The review places emphasis on informed consent and on the pre-operative teaching that reduces later problems.

Read this section on ncbi.nlm.nih.gov →

Technique or Treatment

StatPearls review section · Last updated 2023-07-18

Two main implant types are described, saline-filled and silicone-filled, both with a silicone outer shell that can be smooth or textured. Implants can sit above or beneath the pectoralis muscle, be reached through inframammary, transaxillary or peri-areolar incisions, and are usually done as an outpatient operation under general anaesthesia.

- Saline implants are inserted empty and filled through a port, typically with a 25 to 50 mL range specified by the manufacturer.
- Saline implants are FDA-approved from age 18 for augmentation and at any age for reconstruction; silicone gel implants are approved from age 22.
- Silicone implants suit patients with thin soft-tissue coverage better, showing less rippling and a softer, more natural feel.
- Texturing prevents rotation in shaped implants and may lower capsular contracture in the subglandular pocket, but not in the submuscular pocket.
- No single placement method is uniformly accepted as best; the choice is made jointly by patient and surgeon.
- The operation generally takes 45 to 90 minutes; most patients return to light work within a week, with full activity taking up to six weeks.

Read this section on ncbi.nlm.nih.gov →

Complications and outcomes

Early and late complications, capsular contracture, implant longevity, reported satisfaction and how the care team reduces risk. (4)

Complications

StatPearls review section · Last updated 2023-07-18

Complication rates are described as low, but no implant is a lifetime device. Early problems include infection, bleeding and sensory change; later problems include malposition, deflation or leak and capsular contracture, which may require further surgery.

- Surgeons commonly estimate a 15 to 20 year lifespan for implants, with no uniform recommendation on routine replacement.
- Early complications listed are infection, scarring, asymmetry, haematoma, seroma, breast pain, poor cosmetic outcome and changes in nipple or breast sensation.
- Later complications listed are implant malposition or displacement, implant deflation or leak, and capsular contracture.
- Capsular contracture is graded 1 to 4 and may require surgical correction or implant removal when severe.
- Staphylococcus epidermidis and Cutibacterium acnes are the microorganisms most commonly isolated in studies of capsular contracture, supporting a suspected biofilm or low-grade infection mechanism.
- A slim possibility of breast implant-associated anaplastic large cell lymphoma (ALCL) is reported, possibly linked to a particular type of implant texturing, although the exact relationship is not yet determined.

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Clinical Significance

StatPearls review section · Last updated 2023-07-18

The procedure remains highly popular, and the review stresses that clinicians treating women with implants should be aware of them, because the implants are relevant to other conditions and to screening.

- The American Society of Plastic Surgeons reported approximately 290,000 augmentation procedures performed in the United States in 2016.
- The vast majority of patients report improved self-esteem and satisfaction with the decision.
- Doctors treating these patients for other conditions should have an awareness of the procedure and of implants in general.

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Enhancing Healthcare Team Outcomes

StatPearls review section · Last updated 2023-07-18

Risk reduction is described as a team effort: informed consent, nurse-led preoperative instruction, smoking cessation, pharmacist medicines checks and early recognition of problems. Outcomes are reported as good, with most patients pleased with the result.

- Many past litigations arose primarily from a lack of informed consent, which the pre-operative visit is meant to address.
- Patients should be instructed to stop smoking and make positive lifestyle changes so results last longer.
- Pharmacists contribute by checking allergies and explaining medicines, which can reduce infection and help pain control.
- Surveys cited in the review indicate about 70 to 80 percent of patients are satisfied and feel the outcome was perfect.

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References and Further Reading

StatPearls reference list · Last updated 2023-07-18

The review cites twelve sources spanning secondary augmentation, transgender breast contouring, anatomical versus round implants, fat grafting, capsular contracture risk factors, cosmetic-tourism infection, subclinical implant infection, outcomes series and long-term follow-up.

- The full reference list, with links to each PubMed record, is reproduced in the references section of this page.
- Every citation can be checked at source through its PMID link.

Read this section on ncbi.nlm.nih.gov →

Extracted data

Key facts from the review

The statements below summarise what the StatPearls review says about breast augmentation. The right-hand column links to the section each statement came from.

Topic What the review says Source section What the procedure is Breast augmentation is a surgical procedure in which breast size is enhanced, either by placing an implant or, less commonly, through fat transfer. Introduction How common it is The review describes augmentation as one of the most commonly performed plastic surgery procedures in the United States, and the American Society of Plastic Surgeons reported about 290,000 procedures in 2016. Clinical Significance Silicone implant history The FDA restricted silicone-filled implants for cosmetic enhancement from 1992, and allowed them again without clinical-study enrolment in 2006 after studies failed to confirm disease links. Introduction Implant monitoring The FDA recommends MRI screening every two years for patients with silicone implants to detect subclinical leaks, but the review notes that

patients and physicians do not always follow this and that MRI can be falsely positive. Introduction Implant longevity No implant is a lifetime device: surgeons commonly estimate a 15 to 20 year lifespan, and there is no uniform recommendation on routine replacement at a set interval. Complications Gel cohesion Current implants use more cohesive, or 'gummy', gel than earlier generations, which may lower rupture rates, but the review states that all implants will eventually rupture or break. Introduction Breast anatomy The breast sits on the anterior chest wall below the clavicle, bounded medially by the sternum, laterally by the latissimus dorsi and inferiorly by the inframammary fold, with the pectoralis major and minor behind it. Anatomy and Physiology Blood and nerve supply Blood supply comes from the subdermal plexus with contributions from the internal mammary, external mammary and intercostal perforators; sensation is mainly from the third to fifth intercostal nerves. Anatomy and Physiology Indications Indications include reconstruction after mastectomy, developmental disorders such as hypomastia, including involution after pregnancy, and elective enhancement of self-image. Indications Patient satisfaction The review states that the vast majority of women surveyed after surgery would undergo the operation again, and surveys cited indicate about 70 to 80 percent of patients are satisfied with the outcome. Indications Contraindications Active breast infection, active cancer and pregnancy rule out surgery; autoimmune disease history, current radiotherapy, unstable medical conditions and known silicone sensitivity are also listed. Contraindications Patient selection Psychological instability or unrealistic expectations are given as reasons not to operate, alongside the medical contraindications. Contraindications Saline implants Saline implants are inserted empty and inflated through a port, usually with a 25 to 50 mL fill range set by the manufacturer, and are FDA-approved from age 18 for augmentation. Technique or Treatment Silicone implants Silicone gel implants are FDA-approved for patients aged 22 and over, though surgeons may use them off-label below 22, and they suit thin patients with little soft-tissue coverage better. Technique or Treatment Surface texture Textured shells help prevent rotation of shaped implants and may lower capsular contracture when the implant is placed in the subglandular pocket, but not in the submuscular pocket. Technique or Treatment Implant placement Implants can be placed above the muscle (subglandular) or beneath it (submuscular); no single placement is accepted as uniformly best and the choice is made jointly with the surgeon. Technique or Treatment Incision approaches Common access incisions are the inframammary crease, the transaxillary route and the peri-areolar route. Technique or Treatment Anaesthesia and duration General anaesthesia is most common, although sedation or local anaesthesia can be used in selected cases, and the operation is an outpatient procedure taking 45 to 90 minutes. Technique or Treatment Recovery Recovery is usually rapid, with a return to light work and activity within a week and a return to full activity that may take up to six weeks. Technique or Treatment Theatre team The operating team normally consists of the surgeon, a scrubbed assistant, an anaesthesiologist and a circulating nurse. Personnel No-touch technique An implant sleeve may be used as part of a no-touch technique that minimises handling of the implant, to reduce implant trauma and bacterial contamination. Equipment Early complications Early postoperative complications listed are infection, scarring, asymmetry, haematoma, seroma, breast pain, poor cosmetic outcome and changes in nipple or breast sensation. Complications Late complications Later complications listed are implant malposition or displacement, implant deflation or leak, and capsular contracture, which is tightening of the tissue capsule around the implant. Complications Capsular contracture Capsular contracture is graded 1 to 4 and may need surgical correction or implant removal; a low-grade infection or biofilm is suspected as a cause, with Staphylococcus epidermidis and Cutibacterium acnes most commonly isolated. Complications Implant-associated lymphoma The review reports a slim possibility that women with breast implants are more likely to be diagnosed with anaplastic large cell lymphoma (ALCL), possibly associated with a certain type of texturing, although the exact relationship has not been determined. Complications Reducing complications Nurse-led preoperative instruction, stopping smoking, pharmacist allergy and medicines checks, and interprofessional follow-up are described as ways to lower complication rates, with earlier recognition improving outcomes. Enhancing Healthcare Team Outcomes

Statements are paraphrased from the StatPearls review NBK482206, last updated 2023-07-18. Verify against the linked NCBI Bookshelf section before relying on any point, and note that clinical practice and regulatory positions change over time.

Source, citation and licence

Fardo D, Sequeira Campos MB, Pensler JM. Breast Augmentation. [Updated 2023 Jul 18]. In: StatPearls [Internet]. Treasure Island (FL): StatPearls Publishing; 2023 Jan-. The review is distributed by its publishers under a CC BY-NC-ND 4.0 licence; this page stores paraphrased summaries, extracted facts and links to the freely readable original on ncbi.nlm.nih.gov.

Descriptions of indications, approvals and monitoring reflect United States practice at the time of writing; standards, product approvals and screening advice differ between countries.

For published studies rather than a single clinical review, see the PMC research library . For regulator guidance, see the FDA information library and the NHS information library . For academic patient education, see the Cleveland Clinic information library . For our own treatment guides, see breast augmentation and the evidence guide .

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All 12 sources from the review's reference list, each linking to its PubMed record.

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Wikipedia Breast Augmentation Data: Implants, Procedures & Risks | Breast Enlargement Guide

Page: /research/wikipedia-breast-augmentation/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Source library

Wikipedia: Breast Augmentation Data

Everything the English Wikipedia article “Breast augmentation” covers — prostheses, surgical technique, recovery, complications, fat grafting and the patient — captured at a fixed revision and linked to the original.

This library captures the content of the English Wikipedia article “Breast augmentation” (also called breast enlargement) — its definition and three therapeutic purposes, the four implant types, the five incision routes and four pocket placements, recovery and complication data, fat-graft technique, the breastfeeding and toxicity sections, the psychology findings, 23 extracted facts and the key references. Every entry links back to the original article so wording and later edits can be checked at source.

What this library is — and what it is not

Wikipedia is a collaboratively edited, open encyclopedia. It is a starting point for orientation, not medical guidance: its articles are not written or reviewed by a clinician, and their content can change between visits. Nothing here is a recommendation of a procedure, implant or surgeon, and individual suitability always requires assessment by a qualified healthcare professional.

The data on this page was captured from revision 1381092199 of the article, last edited on 20 October 2026. Wikipedia articles change over time — always read the current version before relying on any point.

Coverage

What the Wikipedia article covers

A long, technical article with 36 sections spanning prostheses, operative technique, complications, fat grafting and the patient.

Implant prostheses

Saline, silicone-gel (five generations), alternative-composition and structured implants, plus their commercial history.

Operative technique

Three therapeutic purposes, five incision routes (inframammary to transabdominal) and four pocket placements.

Recovery and complications

Healing timelines, complication rates, rupture and detection, capsular contracture, revision surgery and systemic sickness.

Fat-graft augmentation

Autologous fat injection: indications, harvesting and refinement, external tissue expansion, post-mastectomy reconstruction and limits.

Function and safety

Breastfeeding with implants, breast-filler toxicity and the platinum catalyst used in silicone gel.

The patient

Pre-operative psychology, reported satisfaction and self-esteem gains, bodybuilders, and the mental-health findings with their caveats.

Indications

The three therapeutic purposes

The article frames every prosthetic-breast placement under one of three purposes.

Purpose What the article says Primary reconstruction Repairing chest and breast tissue damaged by breast cancer, blunt or penetrating trauma or blast injury, and correcting developmental defects of the chest such as tuberous breast deformity. Revision and reconstruction Correcting complications or an unsatisfactory outcome of a previous mammoplasty procedure. Primary augmentation Electively enhancing the aesthetics of the bust — cup size, form and texture of the breasts.
Condensed from the article's lead and "Breast surgery procedures" section.

All sections captured from the article

Grouped by topic. Each entry gives a short summary, the main extracted points and a direct link to that section of the Wikipedia article.

Breast-implant prostheses

The four types of prosthesis the article describes, the history of each technology and the five generations of silicone-gel design.

Saline breast prosthesis

Article section

The history, surgical handling and cosmetic trade-offs of salt-water-filled implants.

- First commercial saline prosthesis presented by Laboratoires Arion (France) in 1964.
- Inserted empty through a short incision, then filled with saline and contoured for symmetry.
- Good-to-excellent size and contour results, but rippling can show in women with little breast tissue.
- With ample breast tissue and submuscular placement, results match those of silicone-gel implants.

Read this section on Wikipedia →

Silicone-gel breast prosthesis

Article section

Five generations of silicone-gel implant design, from the 1963 Cronin–Gerow teardrop to today's cohesive gel.

- Invented in 1961 by Frank Gerow and Thomas Cronin; manufactured by Dow Corning; first used in 1962.
- Second-generation thin shells proved fragile, with higher rupture, leakage and capsular contracture rates.
- Polyurethane coating reduced contracture but was dropped in the U.S. over TDA, a carcinogenic breakdown product; still used in Europe and South America.
- Fifth-generation semi-solid cohesive gel reduces leakage and migration, with lower contracture and rupture rates in 2004–2005 studies.

Read this section on Wikipedia →

Alternative-composition breast prosthesis

Article section

Historic fillers — soy oil, polypropylene string, cartilage, wool, rubber, Teflon-silicone — described as harmful and discontinued.

- Listed as the third category of prosthetic breast.
- The article describes these filler substances as harmful to the woman's body.

Read this section on Wikipedia →

Structured breast prosthesis

Article section

Nested saline-filled shells approved in 2014 in the U.S. and Canada; now discontinued.

- Approved by the FDA (U.S.) and Health Canada in 2014.
- Three nested silicone-rubber shells; saline fills the two spaces between them.
- Inserted folded and empty, then filled with saline in the pocket.

Read this section on Wikipedia →

Surgical technique

The three therapeutic purposes, the five surgical incisions and the four implant-pocket placements.

Types of surgical incision

Article section

Five incision routes for placing a prosthesis: inframammary, periareolar, transaxillary, transumbilical and transabdominal.

- Inframammary gives the best access; periareolar is short but raises contracture and duct/nerve risks.
- Transaxillary, TUBA and TABA routes avoid scarring the breast skin.
- Incision choice shapes scar visibility, access, and breastfeeding risk.

Read this section on Wikipedia →

Implant-pocket placement

Article section

Four placements relative to the pectoralis major muscle: subglandular, subfascial, subpectoral (dual plane) and submuscular.

- Subglandular is closest to the natural plane but more prone to rippling in thin tissue.
- Dual-plane (subpectoral) covers the upper implant with muscle while letting the lower half expand — with some risk of animation deformity.
- Submuscular places the implant fully under the muscle without releasing its lower origin.
- A 2026 series of 198 dual-plane II patients reported 1.0% revisions for superior malposition and 0.5% delayed mastopexy.

Read this section on Wikipedia →

Recovery and complications

Post-surgical recovery, general medical complications, implant rupture, capsular contracture, revision surgery and systemic sickness.

Post-surgical recovery

Article section

Scar healing, return to normal activities and exercise limits after augmentation mammoplasty.

- Scars heal at about 6 weeks and fade over several months, depending on skin type.
- Most patients resume normal daily activities at about 1 week after surgery.
- Submuscular placement means longer convalescence and more pain, because chest muscles were cut.
- No exercise or strenuous activity for about six weeks; gentle arm movement is encouraged early.

Read this section on Wikipedia →

Medical complications

Article section

Surgical risks and breast-specific complications, including reported rates and the 2019 Allergan recall.

- Usual surgical risks: adverse anaesthetic reaction, haematoma, seroma and wound infection.
- Breast-specific complications: pain, altered sensation, wrinkling or asymmetry, skin thinning, impeded breastfeeding and symmastia.
- Implant surgery and tissue-expander complications occur in about 6–7% of patients.
- By 10 years, about 20% of aesthetic and 50% of reconstruction patients needed explantation of a failed prosthesis.
- In 2019 the FDA recalled all Allergan Biocell textured implants over BIA-ALCL, a lymphoma linked to their surface.

Read this section on Wikipedia →

Rupture of the prosthetic breast

Article section

How the body's fibrous capsule can compress an implant until it ruptures, intracapsularly or extracapsularly, and how rupture is detected.

- The immune system encases any implant in a collagen capsule; thickening can compress and rupture it.
- Intracapsular rupture keeps filler inside the capsule; extracapsular rupture lets it migrate into the thorax, causing granulomas or lymphadenopathy.
- Rupture is often unnoticed at first because saline and silicone gel are biologically inert.
- Causes include surgical damage, later thoracic surgery, shell degradation, trauma and mechanical pressure such as capsular contracture or mammography.
- Manual examination detects about 30% of ruptures; MRI detects about 86%.
- The FDA advises an MRI at 3 years after surgery, then every two years for suspected rupture.
- Reported saline rates: 3–5% rupture at 3 years and 7–10% at 10 years; ~10–13% overfilling cut the 8-year deflation rate to 1.83%.

Read this section on Wikipedia →

Capsular contracture

Article section

The scar capsule tightening around an implant until it deforms or breaks the device — causes, prevention and treatment, plus revision surgery.

- Causes include bacterial contamination, shell rupture, filler leakage and haematoma.
- Lower contracture rates are associated with submuscular placement, textured surfaces, minimal implant handling and antibiotic irrigation.
- Treatment ranges from open capsulotomy with implant replacement to non-surgical options (massage, ultrasound, leukotriene inhibitors, pulsed electromagnetic field therapy).
- Indications for revision surgery: complications, capsular contracture, shell rupture, gel leakage and rupture-and-

deflation.

Read this section on Wikipedia →

Systemic sickness

Article section

Study evidence on whether silicone-gel implants cause systemic or autoimmune disease — consistently no causal link found.

- Reviews since the 1990s report no causal relation to systemic or autoimmune disease, though some women report neurological and rheumatological symptoms.
- A 2004 Danish study found no higher autoimmune incidence than the general population.
- European and North American cohorts found augmentation patients generally healthier than the general population, with lower standardized mortality ratios.
- A fibromyalgia association reported in 2001 was criticised as methodologically flawed; the FDA found no support in the literature.
- A 25,000-woman Canadian mortality study (2006) found breast prostheses do not directly increase mortality.

Read this section on Wikipedia →

Fat-graft augmentation

Autologous fat grafting: indications, injection techniques, external tissue expansion, post-mastectomy reconstruction and technical limits.

Fat-graft breast augmentation

Article section

Autologous fat injection for reconstruction, congenital defect correction and aesthetic enhancement — history, evidence and risks.

- Indications: post-mastectomy reconstruction, trauma and explantation deformity; micromastia, tuberous breast deformity and Poland syndrome; and primary aesthetic augmentation.
- Developed by Melvin Bircoll; in 1987 Eduardo Krulig injected grafts with a syringe and blunt needle using a disposable fat trap.
- Up to about 300 ml of body fat can be injected in three equal-volume injections across subpectoral, intrapectoral and submammary spaces.
- Risks: fat necrosis, metastatic calcification, cysts and palpable lumps.
- Adding adipose-derived stem cells raised graft success rates in a 2007 40-patient study.

Read this section on Wikipedia →

Fat grafting techniques

Article section

Harvesting, centrifugal refinement, blunt-cannula injection in small aliquots, and how much of the graft survives.

- Only about 25–50% of transferred fat survives at one year, so compensatory over-filling is used.
- Small-aliquot blunt-cannula injection maximises contact with vascularised tissue and minimises fat necrosis.
- Contours are sculpted by layering grafts at different depths — from the chest wall to just under the skin.
- Operating time: about 2 hours for the first 100 cm^3, then about 45 minutes per extra 100 cm^3.
- Fat grafted around an implant can hide device edges and wrinkles and soften the capsule.

Read this section on Wikipedia →

External tissue expansion

Article section

Vacuum pre-expansion of the recipient site before fat grafting, with the reported volume gains from two studies.

- In a 17-woman study, a bra-like vacuum system applied 20 mmHg for 10–12 hours daily over 10 weeks.
- Average volume gain at week 10 was about 98%, with a stable long-term increase of 55% (range 15–115%).
- A 2010 study of 25 patients (46 breasts) reported significant volume increases of 60–200% at six months by MRI, with no oil cysts or abnormality.
- MRI and 3-D volumetric imaging are used to monitor graft viability and volume maintenance.

Read this section on Wikipedia →

Post-mastectomy procedures

Article section

Implant and flap reconstruction options, and fat-graft reconstruction as a non-implant alternative.

- Surgical options include implants, TRAM flap, latissimus myocutaneous flap and DIEP flap.
- Fat-graft reconstruction begins with 3–5 weeks of external vacuum expansion, then serial grafting sessions over about three months.
- Some reconstructions begin in the operating room at the mastectomy itself, seeding fat into the recipient site.
- Reported outcomes: natural form and near-normal sensation, with resumption of normal activities at about 3 days.

Read this section on Wikipedia →

Tissue engineering: the breast mound and explantation deformity

Article section

Pre-expanded, fat-seeded breast mounds after mastectomy, and using fat grafts to correct empty implant pockets after explantation.

- A 2010 series: 33 women (47 breasts) pre-expanded for 10 hours daily over 10–30 days, then grafted with 150–600 ml per session across 2–5 outpatient procedures.
- Stable reconstructed volumes of 300–600 ml per breast at six months, with benign oil cysts and rare complications (pneumothorax, transient cysts).
- After implant removal, fat grafts replace about 50% of the removed volume as permanent augmentation.
- Fat cannot survive in an empty implant pocket — it must be injected into a vascularised tissue matrix.

Read this section on Wikipedia →

Fat-graft augmentation outcomes and limitations

Article section

Reported volume gains from fat grafting and why it cannot replace the volumes implants deliver.

- A 2010 study of 25 patients: mean breast volume increased 1.2× (60–80%, about 250 cm^3 per breast) at six months, confirmed by quantitative MRI.
- Graft volumes of 220–650 cm^3 per breast; mean operating time 2 hours; no infection, cyst, seroma, haematoma or necrosis reported.
- Structural fat grafting gives precise contour control but cannot achieve the large global volume increases implants provide.
- In one session, even patients with ample donor fat gained about one bra cup size.
- Radiotherapy of an implant-augmented breast sharply raises complication rates (contracture, infection, extrusion, poor cosmetic outcome).

Read this section on Wikipedia →

Function and safety

Breastfeeding with implants, breast-filler toxicity and platinum toxicity.

Breastfeeding and the prosthetic breast

Article section

Whether implants interfere with breastfeeding, and which surgical choices preserve lactation.

- Women with implants can breastfeed, but surgery that cuts into the nipple-areola complex can damage ducts or nerves.
- IMF, transaxillary and transumbilical approaches avoid the nipple-areola complex.
- Subglandular placement and oversized implants interfere most with lactation; small implants in a submuscular pocket interfere least.
- Filler leakage into milk is considered a minimal risk: silicone is indigestible and saline digestible.
- A 1996 study found no causal link between prostheses and impaired breastfeeding function.

Read this section on Wikipedia →

Breast-filler toxicity and platinum toxicity

Article section

Theoretical filler transfer to milk, and the platinum catalyst used to make silicone gel — both reviewed as minimal risks.

- Breast-filler toxicity is described as a minimal risk to a nursing infant because silicone is indigestible and saline digestible.
- Platinum (Pt 78) is the catalyst that turns liquid silicone oil into gel; trace platinum could theoretically accumulate in bone marrow.
- The FDA found the platinum in modern implants is not ionised and poses no biological risk, with no reported cases of trace-platinum toxicity.

Read this section on Wikipedia →

The patient

Psychology before surgery, outcomes reported afterwards, women bodybuilders and mental-health findings.

Psychology of breast-augmentation patients

Article section

Mental-health characteristics of women seeking augmentation, and the outcomes they report after surgery.

- Pre-operative studies report more prior psychotherapy, low self-esteem, depression and body dysmorphia than in the general population.
- Post-operative surveys report improved physical health, appearance, social life, confidence, self-esteem and sexual functioning.
- Most women report long-term satisfaction with their implants, even after revision surgery.
- About 8% of Danish augmentation patients had a pre-operative psychiatric hospitalization history.

Read this section on Wikipedia →

Mental health and mortality findings

Article section

Suicide and mortality findings, and measured self-esteem gains — with the article's conclusion on causation.

- A 2007 longitudinal study found higher suicide rates among women with implants over time, alongside higher

alcohol- and drug-related death risk.

- The article's conclusion: surgery does not increase suicide risk — it is the psychologically affected woman who is more likely to seek augmentation.
- A 2007 study reported self-esteem gains of 20.7–24.9 points on the 30-point Rosenberg scale after surgery.
- Plastic surgeons are advised to assess mental health before operating, to judge whether implants can genuinely help.

Read this section on Wikipedia →

Surgical technique

Five types of surgical incision

How the prosthesis reaches the implant pocket — and what each route means for scars, access and breastfeeding.

Incision What the article says Inframammary incision A cut at the inframammary fold (the bottom border of the breast) giving maximal access to the interior of the breast for precise pocket creation and implant placement. Scarring can be noticeable depending on skin type. Periareolar incision A short (~5 cm) cut along the lower half of the areola's outer border. The narrow access makes large silicone implants harder to place, tends toward higher capsular contracture rates long term, and risks damaging milk ducts and nerves of the nipple-areola complex, which can impede breastfeeding. It also permits a concurrent breast lift. Transaxillary incision A cut in the armpit with a tunnel under the skin to the breast pocket, done bluntly or with an endoscope. It avoids scarring the breast skin; the challenge is judging the ideal implant position for symmetry. Transumbilical incision (TUBA) An endoscopic approach through the navel, tunneling upward under the abdominal skin to place a saline implant. It avoids scarring the breast skin. Transabdominal incision (TABA) An endoscopic approach through the navel tunneling up to the breast pocket, performed while the patient simultaneously undergoes an abdominoplasty.
Condensed from the “Types of surgical incision” section. The incision route is chosen with the surgeon according to anatomy, implant type and goals.

Surgical technique

Four implant-pocket placements

Placements described in anatomical relation to the pectoralis major muscle.

Placement What the article says Subglandular The implant sits between the breast tissue and the pectoralis major muscle (retromammary space), closest to the natural breast plane and usually the most aesthetic — but more prone to visible rippling in women with little soft tissue. Subfascial The implant is placed beneath the fascia covering the pectoralis major muscle, to add size and volume for later contouring. Subpectoral (dual plane) The implant is placed beneath the pectoralis major muscle after partially releasing its lower attachments: the upper half of the implant is covered by muscle, the lower half sits in the subglandular plane. It maximizes upper coverage and lower expansion, at some risk of animation deformity (the implant moving with the muscle). Submuscular The implant is placed entirely beneath the pectoralis major muscle without cutting its lower origin; total coverage can be completed by releasing and reattaching lateral chest-wall muscles (serratus or pectoralis minor).
Condensed from the “Implant-pocket placement” section. Placement affects coverage, rippling risk, recovery and how the implant moves.

Implant types

The four prosthesis types

The four categories of breast prosthesis the article describes, two of which are discontinued.

Prosthesis Category Summary Detail Saline breast prosthesis Implant type A silicone-elastic shell filled with sterile saline solution in the operating room. First sold commercially by Laboratoires Arion of France in 1964.

The empty shell is inserted through a short incision, then filled with saline and contoured for symmetry. Results are good to excellent, but rippling and wrinkling of the skin can show, especially in women with little breast tissue; with ample tissue and submuscular placement, results resemble those of silicone-gel implants. Silicone-gel breast prosthesis Implant type A shell filled with viscous silicone gel; the technology has passed through five generations of design. Invented in 1961 by Frank Gerow and Thomas Cronin, made by Dow Corning, and first used in augmentation in 1962. Generation 1 (1963) was a teardrop silicone-rubber sack with a Dacron fastener patch. Generation 2 (1970s) used thin shells and low-cohesion gel — fragile, with higher rupture, leakage and capsular contracture rates that prompted class-action lawsuits; polyurethane-coated variants reduced contracture but were dropped in the U.S. over carcinogenic TDA by-products (they remain in use in Europe and South America); double-lumen designs combined gel and saline with post-operative volume adjustability. Generations 3 and 4 (1980s) added elastomer coatings to reduce gel bleed, with shaped, tapered and round textured or smooth models. Generation 5 (1990s) uses semi-solid cohesive gel that resists leakage and migration, with studies reporting lower capsular contracture and rupture rates than earlier generations. Alternative-composition breast prosthesis Implant type (discontinued) Fillers such as soy oil, polypropylene string, ox cartilage, Terylene wool, ground rubber, silastic or Teflon-silicone — now discontinued. The article lists these fillers as substances harmful to the body and no longer in use. Structured breast prosthesis Implant type (discontinued) Nested silicone-rubber shells with saline between them, approved by the FDA and Health Canada in 2014; now discontinued. Three nested shells support the upper half of the breast and the two spaces between them are filled with saline, combining saline and silicone technologies. The folded empty device is inserted, then filled with saline and the incision sutured.
Condensed from the “Breast-augmentation prostheses” section. Implant availability differs by country and regulator.

Extracted data

Key facts from the article

The statements below condense what the Wikipedia article says about breast augmentation. The right-hand column links to the source article.

Topic What the article says Source Definition Breast augmentation (also breast enlargement) is a plastic surgery procedure placing a breast implant or fat graft on the thorax to increase breast size, correct congenital defects and restore anatomic proportion. Breast augmentation Three purposes Prosthetic breasts are placed for three therapeutic reasons: primary reconstruction after cancer, trauma or developmental defect; revision of a previous mammoplasty; and primary aesthetic augmentation. Breast augmentation Fat-graft gain A fat-graft augmentation yields a modest size increase — usually about one bra cup size — because the body resorbs much of the injected fat. Breast augmentation Saline history The first commercial saline-filled breast prosthesis was presented by Laboratoires Arion of France in 1964. Breast augmentation Silicone history The silicone-gel implant was invented in 1961 by Frank Gerow and Thomas Cronin, made by Dow Corning, and first used for augmentation in 1962; its technology spans five generations. Breast augmentation Structured implants Structured (nested-shell) breast prostheses were approved by the FDA and Health Canada in 2014; both structured and alternative-composition prostheses are now discontinued. Breast augmentation Incisions Five surgical incision routes are used: inframammary, periareolar, transaxillary, transumbilical (TUBA) and transabdominal (TABA). Breast augmentation Placements Four implant-pocket placements are described: subglandular, subfascial, subpectoral (dual plane) and submuscular. Breast augmentation Recovery Scars heal at about 6 weeks, most patients resume normal activities at about 1 week, and strenuous exercise is avoided for about 6 weeks; submuscular placement means a longer, more painful recovery. Breast augmentation Complication rate Complications from implant surgery and tissue expanders occur in approximately 6–7% of breast-augmentation patients. Breast augmentation Explantation at 10 years About 20% of women with aesthetic implants and 50% of reconstruction patients required explantation of a failed prosthesis at the 10-year mark. Breast augmentation BIA-ALCL recall In 2019 the FDA recalled all Allergan Biocell textured breast prostheses after a causal link was identified with BIA-ALCL, an immune-system cancer. Breast augmentation Rupture detection Manual examination detects only about 30% of implant ruptures, while MRI detects about 86%; the FDA recommends screening MRI at 3 years post-surgery and every 2 years thereafter for suspected rupture. Breast augmentation Saline rupture rates Reported saline rupture-and-deflation rates are 3–5% at 3 years and 7–10% at 10 years; overfilling implants by about 10–13% reduced the 8-year deflation rate to 1.83%. Breast augmentation Revision rates Revision (re-operation) rates were reported as 20% at 3 years and 3.0% at

7 years in the article's cited Tebbetts data. Breast augmentation Systemic disease Reviews and longitudinal studies since the 1990s consistently report no causal relation between silicone-gel implants and systemic or autoimmune disease. Breast augmentation Fat survival Only about 25–50% of a fat graft survives at one year, so grafts are deliberately over-filled; retained contours typically last 18–24 months. Breast augmentation Fat-graft volume gain A 2010 clinical-radiological study reported a mean breast-volume increase of 1.2× (60–80%, about 250 cm^3 per breast) at 6 months after fat grafting, confirmed by quantitative MRI. Breast augmentation External expansion External vacuum pre-expansion (20 mmHg, 10–12 hours daily for 10 weeks) produced an average 98% volume gain at week 10, settling to a stable 55% long-term increase (range 15–115%). Breast augmentation Breastfeeding Small implants in a submuscular pocket interfere least with breastfeeding; subglandular placement and oversized implants interfere most, and IMF/TABA/TUBA approaches avoid the nipple-areola complex. Breast augmentation Self-esteem A 2007 study reported post-operative self-esteem increases of 20.7–24.9 points on the 30-point Rosenberg scale, alongside reports of improved confidence and sexual functioning. Breast augmentation Mental-health caution The article notes higher pre-operative rates of depression, low self-esteem and body dysmorphia among augmentation seekers, and concludes that underlying psychological factors — not the surgery itself — explain the suicide-risk correlation reported in longitudinal studies. Breast augmentation Radiotherapy Radiotherapy of an implant-augmented breast much increases complication rates — capsular contracture, infection, extrusion and poor cosmetic outcome — which can preclude conservative breast surgery. Breast augmentation
Statements are paraphrased from the Wikipedia article. Wikipedia is open content that anyone can edit; verify against the linked article revision before relying on any point.

Key references cited by the article

A selection of the most load-bearing citations from the article’s 133-reference list — the studies and regulator notices behind its rates and findings — with DOI and PubMed links where available.

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- Effects of Breast Augmentation Mammoplasty on Self-Esteem and Sexuality: A Quantitative Analysis (2007) (cited for Rosenberg self-esteem gains of 20.7–24.9 points). PMID 17957218

See the full reference list in the article →

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Wikipedia Breast Enlargement: Methods, Facts & References | Breast Enlargement Guide

Page: /research/wikipedia-breast-enlargement/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Source library

Wikipedia: Breast Enlargement Methods & Data

Everything the English Wikipedia article “Breast enlargement” covers — definition, methods, extracted facts and references — captured at a fixed revision and linked to the original.

This library captures the content of the English Wikipedia article “Breast enlargement” — its definition, the four methods it lists as actual or potential ways to achieve larger breasts, the one method it lists as ineffective, the facts extracted from the text and all seven of its references. Every entry links back to the original article so the wording and any later edits can be checked at source.

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The data on this page was captured from revision 1289906330 of the article, last edited on 11 May 2025. Wikipedia articles change over time — always read the current version before relying on any point.

Coverage

What the Wikipedia article covers

A short article with a definition, four methods, one ineffective method, a see-also list and seven references.

Definition and context

Breast enlargement as natural mammoplasia or active intervention, and why breasts matter to self-image, femininity and sexual attractiveness.

Surgical method

Breast augmentation with implants or fat transfer, listed as an actual and potential method and linked to the augmentation articles.

Hormonal method

Pharmacological or hormonal enhancement with medications such as estrogen or breast enlargement supplements, with three supporting citations.

Food energy and KLF14

Increasing food energy intake to add adipose tissue near the breast area, with distribution between individuals controlled by the KLF14 gene.

Mechanical method

Mechanical breast enhancement, cited to a 2007 study of the Brava external tissue expander.

Ineffective methods

The Mark Eden bust developer, named and described as a fraudulently marketed method and device.

The article in brief

Breast enlargement is the enlargement of the breasts. It may occur naturally as in mammoplasia or may occur artificially through active intervention. Many women regard their breasts, which are female secondary sex characteristics, as important to their sexual attractiveness, as a sign of femininity that is important to their sense of self. Due to this, when a woman considers her breasts deficient in some respect, she might choose to engage in some activity intended to enhance them.

Mammoplasia. The article names natural breast enlargement as mammoplasia, distinguishing it from enlargement produced by active intervention.

Methods listed

Methods the article lists to achieve larger breasts

The four actual and potential methods named in the article, with the wording condensed from the source. The right-hand column points to the article’s own references.

Method Category What the article says Refs Surgical breast augmentation Surgical The article lists surgical breast augmentation as an actual and potential method to achieve larger breasts, naming two routes: breast implants and fat transfer (fat grafting). It links to the breast augmentation and breast implant articles for the operative detail, cost and risk information held there. — Pharmacological or hormonal breast enhancement Hormonal The article lists pharmacological or hormonal breast enhancement through administration of medications such as estrogen or breast enlargement supplements. Three of the article's seven citations support this method: two clinical texts on hormone therapy and breast cancer metastasis, and a 1998 study on hormonal breast augmentation and insulin-like growth factor-I (IGF-I). [2] [3] [4] Increasing food energy intake Weight and body composition The article states that by increasing food energy intake, more adipose tissue will be created, part of which will consist of adipose tissue located near the breast area. The amount added to the breasts varies from person to person and is controlled by the KLF14 gene. The article notes that additional modification of the KLF14 gene can thus make sure even more of the adipose tissue increase will occur at the breasts. [5] [6] Mechanical breast enhancement Mechanical / external The article lists mechanical breast enhancement as a potential method, supported by a single citation: a 2007 Plastic and Reconstructive Surgery study evaluating the Brava external tissue expander and asking whether breast enlargement without surgery is a reality. [7] Statements are condensed from the Wikipedia article “Breast enlargement” (revision 1289906330). Inclusion in the article is not evidence of safety or effectiveness — check the linked references and discuss any option with a qualified clinician.

Ineffective methods

Methods the article lists as ineffective

The article separates methods it describes as actual or potential from those it describes as ineffective.

Method Category What the article says Refs Mark Eden bust developer Ineffective The article places the Mark Eden bust developer in its list of ineffective methods to achieve larger breasts, describing it as a fraudulently marketed method and device. —
The article names only one ineffective method. Absence of other entries from this list does not mean a method is effective — evidence for each option has to be assessed separately.

Extracted data

Key facts from the article

The statements below condense what the Wikipedia article says about breast enlargement. The right-hand column links to the source article.

Topic What the article says Source Definition Breast enlargement is the enlargement of the breasts; it may occur naturally as in mammoplasia or artificially through active intervention. Breast enlargement Why it is sought Many women regard their breasts, a female secondary sex characteristic, as important to sexual attractiveness and as a sign of femininity, so a woman who considers her breasts deficient may choose an activity intended to enhance them. Breast enlargement Surgical method Surgical breast augmentation — with breast implants or fat transfer — is listed as an actual and potential method to achieve larger breasts. Breast enlargement Hormonal method Pharmacological or hormonal enhancement through medications such as estrogen or breast enlargement supplements is listed, with citations to a hormone-therapy text, a breast cancer metastasis text and a 1998 study of IGF-I in hormonal breast augmentation. Breast enlargement Food energy intake Increasing food energy intake creates more adipose tissue, part of it located near the breast area; the amount added to the breasts varies between people. Breast enlargement KLF14 gene The amount of adipose tissue added to the breasts is controlled by the KLF14 gene, so modification of KLF14 could direct more of the tissue increase to the breasts. Breast enlargement Mechanical method Mechanical breast enhancement is listed as a potential method, cited to a 2007 study of the Brava external tissue expander. Breast enlargement Ineffective method The Mark Eden bust developer is named as an ineffective method — a fraudulently marketed method and device. Breast enlargement Article scope The article lists four actual or potential methods, one ineffective method and seven references, and is cross-listed in the Wikipedia category 'Breast'. Breast enlargement Statements are paraphrased from the Wikipedia article. Wikipedia is open content that anyone can edit; verify against the linked article revision before relying on any point.

References cited by the article

All seven references from revision 1289906330, with DOI, PubMed and publisher links where the article provides them.

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See also

The article’s own see-also list, with a link to our guide where one exists on this site.

- Breast augmentation · our guide
- Breast reduction

- Reconstructive surgery
- Penis enlargement

Source, licence and attribution

Text on this page is a condensed, paraphrased summary of the English Wikipedia article “Breast enlargement”, not a verbatim copy. The original article is available under the Creative Commons Attribution-ShareAlike 4.0 licence (CC BY-SA 4.0) ; Wikipedia® is a registered trademark of the Wikimedia Foundation, Inc. This library stores summaries, extracted facts, the reference list and links only.

For the long-form Wikipedia article on the operation itself — implant types, incisions, complications and fat grafting — see the Wikipedia breast augmentation library . For regulator guidance, see the FDA information library and the NHS information library . For academic patient education, see the Cleveland Clinic information library . For published research, see the PMC research library , or read our evidence guide .

Download and reuse

The structured dataset behind this page is available as JSON . It contains the article metadata, overview, the method and ineffective-method lists, extracted facts, the full reference list and the see-also links.

Your journey starts with information

Want to put these methods in context for your own decision?

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Breast Augmentation: Implants & Fat Transfer | Breast Enlargement Guide

Page: /surgical/breast-augmentation/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Surgical · Breast augmentation

A Personalised Approach to Breast Augmentation

Breast augmentation is a surgical procedure designed to increase breast volume or alter breast shape using implants or, in selected cases, the patient's own fat.

It may be considered by people seeking greater fullness, a change in proportions, or restoration of volume following pregnancy or weight change. Treatment planning should account for anatomy, existing breast tissue, skin characteristics, general health, lifestyle, expectations and the individual's willingness to accept surgical risks and future follow-up.

What patients should understand

Before deciding

Implant vs. fat transfer

The difference between implant-based augmentation and fat transfer, and what each can achieve.

Qualified surgeon

Why a qualified and appropriately credentialed plastic surgeon should lead the plan.

Assessment & consent

The need for preoperative assessment and informed consent.

Recovery & scars

Recovery requirements, scar management and follow-up.

Complications

Potential complications and the possibility that revision surgery is needed.

Questions

Breast augmentation FAQs

How long does recovery take?

Recovery varies according to the procedure, surgical technique, individual healing and the clinician's advice. Your surgeon will give you a personalised return-to-activity timeline.

Are the results permanent?

No procedure can promise permanent results. Breast implants are not lifetime devices: additional operations may be needed and complications can occur. Fat transfer results depend on how much transferred fat survives.

How do I know which approach is right for me?

The appropriate option depends on your health, anatomy, expectations, risk tolerance and the evidence for the intended treatment. This should be determined through assessment by an appropriately qualified clinician.

Breast augmentation: data from the American Society of Plastic Surgeons

The figures and lists below summarise patient information published by the American Society of Plastic Surgeons (ASPS), the largest association of board-certified plastic surgeons in the United States. They are reproduced here for education and context. ASPS guidance reflects United States practice and regulation and does not replace assessment by a suitably qualified clinician in your own jurisdiction.

ASPS patient information

What breast augmentation can — and cannot — do

Goal What ASPS patient information says Increase fullness and projection Breast implants or fat transfer can increase the size and projection of the breasts. Improve balance and proportion Augmentation can improve the balance between breast and hip contours. Support body image ASPS notes the procedure may enhance self-image and self-confidence. Breast reconstruction Implants may also be used for breast reconstruction after mastectomy or injury. Severe drooping Augmentation alone does not correct severely drooping breasts; a breast lift may be needed, at the same time or as a separate operation.
ASPS patient information

How the procedure is usually performed

ASPS describes the operation in five broad stages.

Anesthesia

Intravenous sedation or general anaesthesia is used, so the procedure is comfortable throughout.

Incision

Peri-areolar, inframammary fold or axillary incisions are the usual routes; ASPS notes that the belly-button approach carries a higher complication rate.

Placement

The implant is placed either under the pectoral muscle (submuscular) or behind the breast tissue over the muscle (subglandular).

Closure

Layered sutures close the tissue, with sutures, skin adhesive or surgical tape on the skin. Scars fade over time and are affected by genetics, nicotine and infection.

Results

Results are visible immediately, with the final shape settling as swelling resolves.

ASPS patient information

Types of breast implants

Implant What ASPS patient information says Saline Sterile salt water; collapses and is absorbed or expelled if the shell leaks; uniform shape and firmness; FDA-approved from age 18. Structured saline Salt water with an internal structure for a more natural feel. Silicone gel Gel feels more like natural breast tissue; does not collapse if it leaks; regular ultrasound or MRI screening is advised; FDA-approved from age 22. Form-stable (“gummy

bear”) Keeps its shape even if the shell breaks; firmer and thicker; a shaped version gives more bottom projection but can look unusual if it rotates; requires a longer incision. Round Generally a fuller look; higher profiles add more projection; less concern about rotation. Smooth Softest feel and natural movement, but rippling may be felt or seen. Textured Tissue adheres to the surface so it is less likely to move and may reduce capsular contracture, but textured surfaces are where BIA-ALCL occurs most often.

Recovery and results (ASPS patient information)

- Immediately after surgery the breasts are wrapped in gauze, with an elastic bandage or support bra to limit swelling and support healing.
- Patients are monitored in recovery and are usually permitted home about an hour after surgery once stable for discharge.
- The support garment (a bra or elastic band) is worn around the clock as instructed; wound care instructions differ between surgeons and should be followed exactly.
- Acute pain typically subsides after one to five days, but soreness and swelling may persist for a few weeks.
- Final results settle over a few weeks as swelling subsides; incision lines may take several months to a couple of years to fade.
- Breasts change over time with ageing, weight fluctuation, hormones and gravity; some people later choose a lift or an implant exchange.
- Implants are not guaranteed to last a lifetime, so further surgery may eventually be needed.

Risks and safety (ASPS patient information)

ASPS lists the following possible risks of breast augmentation surgery, discussed fully before consent:

- Anaesthesia risks.
- Breast implant-associated anaplastic large cell lymphoma (BIA-ALCL) or other very rare cancers in the capsule, such as breast implant-associated squamous cell carcinoma (BIA-SCC).
- Bleeding or haematoma.
- Changes in nipple or breast sensation.
- Fluid accumulation (seroma).
- Tight scar tissue forming around the implant (capsular contracture).
- Implant leakage or rupture.
- Infection and persistent pain.
- Poor scarring, wrinkling of the skin over the implant, or the implant sitting in the wrong position.
- The possibility that revision surgery will be needed.

For silicone implants, the FDA recommends screening with MRI or ultrasound five to six years after placement and every two to three years thereafter. Risk factors include pregnancy, weight change and the menopause. Routine examinations help confirm that an implant is intact, because a ruptured implant sometimes causes no symptoms.

ASPS patient information

US average costs

Procedure ASPS average surgeon fee Breast augmentation with implants $4,875 Breast augmentation with fat grafting $5,719
ASPS averages cover the surgeon’s fee only and exclude anaesthesia, operating-room facilities, medical tests, post-surgery garments and prescriptions. Most health insurance plans do not cover cosmetic breast augmentation or a later revision.

ASPS 2025 statistics

Breast augmentation at a glance

Measure ASPS 2025 data Year-on-year change Breast augmentation rose about 11% in 2025. Rank Second most common cosmetic surgical procedure, behind liposuction. Among women Narrowly the most popular cosmetic procedure, ahead of liposuction. By age Favourite procedure for ages 36–45; gains across ages 26–65; the 66+ group grew fastest overall (+24%). Preference A shift towards smaller, more proportionate implants was noted. Related trends Breast lifts rose about 8% and breast reductions about 7%; 82% of member surgeons reported GLP-1-related consultation requests.
Figures are from the ASPS 2025 plastic surgery statistics and describe the United States, so they do not represent Indian or regional practice.

Source: American Society of Plastic Surgeons

Summarised from ASPS patient information and statistics pages:

- Breast augmentation
- Candidates
- Types of breast implants
- Procedure steps
- Recovery
- Results
- Risks and safety
- Cost
- 2025 plastic surgery statistics

Source: NCBI Bookshelf clinical review

The clinical background on this page is also drawn from the peer-reviewed StatPearls review Breast Augmentation (Bookshelf ID NBK482206, PMID 29489168), published by the US National Library of Medicine. Its sections on indications, contraindications, implant choices, placement, incisions, recovery, complications and outcomes are indexed in our NCBI Bookshelf breast augmentation library , with links to each section of the original.

Read the full review on ncbi.nlm.nih.gov — it is free to read, and it describes United States practice and regulation at the time of writing.

Your journey starts with information

Plan a personalised breast augmentation consultation

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Breast Filler Removal: Clearing Injected Fillers & Silicone | Breast Enlargement Guide

Page: /surgical/breast-filler-removal/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Surgical · Corrective

Breast Filler Removal: Addressing Injected Fillers and Silicone

Injectable fillers and liquid silicone are not approved for breast enlargement. When they cause pain, migration, lumps or infection, removal surgery may be required.

Fillers are not implants

Dermal fillers approved for facial use must never be assumed suitable for the breasts. Injected silicone and fillers can migrate, form granulomas, cause chronic inflammation and infection, and complicate breast imaging. They are not an equivalent or safer substitute for a breast implant.

People who have had fillers or liquid silicone injected into the breasts may present with pain, hardness, migrating lumps, asymmetry, skin changes or recurrent infection. Removal is corrective surgery: the injected material is not neatly contained like an implant, and it interleaves with breast tissue, so complete removal is not always possible in one operation.

Surgical approaches may include removal of the injected material together with the surrounding affected tissue, staged procedures, and in some cases combining removal with implant placement or a lift once healthy tissue has healed. Imaging (such as ultrasound or mammography) is usually assessed before planning, because injected material can distort images and mask or mimic lesions.

What patients should understand

Key considerations

Removal may be incomplete

Injected material spreads through tissue; complete clearance in one operation cannot be promised.

Imaging first

Breast imaging is typically reviewed before surgery to map the material and assess the tissue.

Staged treatment

More than one operation may be needed, with healing periods in between.

Scarring & shape

Removal can leave contour changes; reconstruction options are discussed case by case.

Pathology

Removed tissue may be sent for histological examination, as routine.

Delayed presentation

Complications can appear years after injection, so history matters at consultation.

Questions

Breast filler removal FAQs

Can all injected material be removed?

Not always. Injected silicone and fillers intermingle with breast tissue rather than sitting in a pocket, so complete removal in a single operation is not always achievable. The surgical aim is to remove as much affected material and tissue as safely possible.

Can implants be placed at the same time as removal?

Sometimes, but staged surgery is often safer when there is chronic inflammation or infection. Your surgeon will advise based on the condition of the tissue.

Do injected fillers affect breast screening?

Injected material can distort imaging and make interpretation harder. It is important to tell radiologists and surgeons about any prior injections, and to keep up with recommended screening.

Your journey starts with information

Discuss filler removal and corrective options with a surgeon

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Breast Lift Surgery (Mastopexy): Shape, Scars & Longevity | Breast Enlargement Guide

Page: /surgical/breast-lift/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Surgical · Mastopexy

Breast Lift (Mastopexy): Restoring Shape

A breast lift raises and reshapes sagging breasts by removing excess skin and repositioning the breast tissue and nipple. It changes shape, not volume.

Sagging (ptosis) develops with ageing, pregnancy, breastfeeding, weight change and gravity. A breast lift addresses it by removing loose skin, reshaping the remaining breast tissue and repositioning the nipple–areola complex higher on the chest. Unlike augmentation, a lift on its own does not add volume — the breast is reshaped using the tissue already present.

The degree of lift required determines the pattern of scars: around the areola alone for mild sagging, plus a vertical line for moderate sagging, or an additional line within the inframammary fold for more significant sagging. Where more fullness is also desired, a lift can be combined with an implant or fat transfer, with the trade-offs of each discussed at consultation.

At a glance

Lift compared with augmentation and reduction

Procedure What it changes Volume Breast lift (mastopexy) Shape and position — raises sagging tissue and repositions the nipple Unchanged Augmentation Volume and fullness — implants or fat transfer Increased Reduction Volume, weight and shape — removes tissue Decreased
Key points

What to understand before deciding

Scars are permanent

Scars fade with time but do not disappear; their pattern depends on the lift required.

Results are long-lasting, not permanent

Pregnancy, weight change and ageing continue to affect the breast over time.

Combination is common

Lift with implant or fat transfer when both shape and volume need change.

Sensation and feeding

Nipple sensation and future breastfeeding may be affected; this is discussed in advance.

Questions

Breast lift FAQs

What is the difference between a breast lift and augmentation?

Augmentation adds volume with an implant or fat; a lift reshapes and raises existing tissue without adding volume. Some people need one, some need both, and the right combination depends on your anatomy and goals.

How long does a breast lift last?

The reshaping is durable, but the breast continues to age. Pregnancy, weight change and gravity gradually affect the result, and some people eventually choose further surgery.

Can a lift be done without visible scars?

No. Every lift technique involves incisions and therefore permanent scars, although they are planned to sit within the areola border or bra line. No procedure can promise scarless results.

Your journey starts with information

Discuss whether a breast lift suits your goals

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Breast Reduction Surgery: Benefits, Techniques & Recovery | Breast Enlargement Guide

Page: /surgical/breast-reduction/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Surgical · Reduction

Breast Reduction Surgery: Relief, Proportion and Recovery

Breast reduction removes skin, fat and glandular tissue to reduce breast size and weight, and repositions the breast into a more proportionate shape.

Very large or heavy breasts can cause persistent neck, shoulder and back pain, shoulder-groove marks from bra straps, skin irritation beneath the breasts, and difficulty with physical activity. Breast reduction addresses both the symptoms and the proportions, by removing excess tissue and reshaping the remaining breast.

The operation is planned around your anatomy, the degree of reduction desired, blood supply to the nipple–areola complex and your overall health. Scarring is an unavoidable part of the procedure; incisions are placed so that scars fall within the bra line or along the areola border where possible. Because breast reduction is performed to relieve documented physical symptoms, it may in some cases be approached through medical insurance — criteria vary and require documentation.

What patients should understand

Key considerations

Symptom relief

Reduction commonly relieves back, neck and shoulder strain caused by breast weight.

Scars are permanent

Incision lines around the areola, vertically, or along the inframammary fold.

Symmetry is approximate

Perfectly equal breasts cannot be promised; some asymmetry is normal.

Recovery period

Several weeks away from heavy lifting and exercise, with gradual return to activity.

Sensation changes

Nipple and breast skin sensation may temporarily or permanently change.

Breastfeeding

Future breastfeeding ability can be affected; this is discussed in advance.

Questions

Breast reduction FAQs

Can breast reduction be combined with a lift?

Breast reduction inherently includes lifting and reshaping the breast. A separate lift is generally not needed, although the specific technique is tailored to each case.

Will the breasts grow back after reduction?

Reduced tissue does not regrow, but substantial weight gain, pregnancy and hormonal changes can alter breast size again. Long-term results depend on stable weight and individual anatomy.

How much tissue is removed?

The amount depends on your anatomy, symptoms and goals, and is planned with your surgeon. The aim is a proportionate, symptom-relieving result rather than a specific cup size, which cannot be guaranteed.

Your journey starts with information

Discuss whether breast reduction could relieve your symptoms

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Breast Revision Surgery: Implant Exchange, Removal & Repair | Breast Enlargement Guide

Page: /surgical/breast-revision/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Surgical · Revision

Breast Revision Surgery: When Implants Need Attention

Breast revision surgery is performed when a previous breast operation needs correction, adjustment or reversal — for example implant exchange, removal, or repair of capsule-related changes.

Breast implants are not lifetime devices. Over time, the capsule of scar tissue that naturally forms around an implant can tighten (capsular contracture), an implant can rupture or change position, or the breasts can change with pregnancy, weight change and ageing. Some people also simply wish to change size. Revision surgery addresses any of these situations.

Revision is often more complex than a first operation: scar tissue, stretched skin, thinned breast tissue and previous implant position all shape what can safely be achieved. Planning should therefore be led by a suitably qualified surgeon who will examine you, review any previous operative records and imaging, and discuss realistic options — which may include implant exchange, removal with or without replacement, capsulectomy, or combining revision with a breast lift.

Common reasons

Why revision surgery is considered

Capsular contracture

Scar tissue tightening around an implant, causing firmness, distortion or discomfort.

Implant rupture or deflation

Loss of implant integrity, confirmed by examination and imaging where indicated.

Position or shape changes

Displacement, rotation, bottoming out or an unnatural appearance over time.

Asymmetry

Differences between sides that developed after the original surgery.

Change of preference

A wish to change size, or to remove implants entirely (explantation).

Tissue changes

Sagging or skin laxity after pregnancy, weight change or ageing.

Revision is individualised

What is possible depends on the remaining breast tissue, skin quality, implant position and your goals. In some cases a good result requires combining implant revision with a breast lift, or accepting that implant removal

alone will leave a less full shape.

Questions

Breast revision FAQs

How long do breast implants last?

There is no fixed lifespan. Implants may remain stable for many years, but additional operations are common over a lifetime of implant use. Regular review and prompt assessment of any new change are recommended.

Can implants be removed without replacement?

Yes — explantation without replacement is possible. The resulting shape depends on how much natural breast tissue remains and the degree of skin laxity; some people choose to combine removal with a lift.

Is revision surgery more complex than first-time surgery?

Often, yes. Existing scar tissue, altered anatomy and thinner breast tissue can make revision longer and less predictable. A thorough consultation, including imaging where indicated, is essential before planning.

Your journey starts with information

Discuss your revision concerns with a qualified surgeon

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Fat Transfer Breast Augmentation: What to Know | Breast Enlargement Guide

Page: /surgical/fat-transfer-breast-augmentation/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Surgical · Autologous fat transfer

Fat Transfer Breast Augmentation: What to Know

Fat transfer breast augmentation removes fat from a donor area using liposuction, processes it, and transfers selected fat to the breasts. It may suit certain patients seeking a relatively modest increase in volume without implants.

The amount of fat that can be transferred safely, and the proportion that remains over time, vary. Some transferred fat may not survive, and more than one procedure may be needed to approach a patient's goals.

Fat transfer data from the American Society of Plastic Surgeons

ASPS describes fat transfer breast augmentation as using liposuction to remove fat from another part of the body and inject it into the breasts, and notes that it is best suited to patients who want a relatively small increase with a natural result. The ASPS average surgeon fee for breast augmentation with fat grafting is $5,719 (United States, surgeon's fee only). ASPS also refers to the technique as breast augmentation with fat grafting, and cites research into mega-volume fat transfer with pre-expansion and into techniques intended to double breast size using the patient's own fat.

ASPS: Fat transfer breast augmentation

Topics to cover

What to discuss at consultation

Candidacy

Who may be an appropriate candidate for fat transfer.

Donor site & liposuction

Donor-site availability and the liposuction component of the procedure.

Realistic expectations

Expected changes and realistic volume limitations.

Recovery

Recovery at both the donor and recipient sites.

Complications

Potential fat necrosis, oil cysts, calcifications, asymmetry and other complications.

Questions

Fat transfer FAQs

Does fat transfer produce permanent results?

Some transferred fat can survive long term, but a proportion may be reabsorbed. The final result varies between individuals, and additional procedures may be considered.

How much volume can be added?

The safe volume and the amount that survives vary individually, so fat transfer is generally associated with a more modest, individualised increase than implant-based augmentation.

Your journey starts with information

Discuss whether fat transfer suits your goals

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Mastectomy & Chest Contouring Surgery: Patient Information | Breast Enlargement Guide

Page: /surgical/mastectomy-chest-surgery/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Surgical · Chest surgery

Mastectomy and Chest Contouring Surgery

Mastectomy removes breast tissue. When performed to create a flatter, more contoured chest, it is often called chest masculinisation or top surgery.

Mastectomy is the surgical removal of breast tissue. It is performed for several reasons, including gender-affirming chest masculinisation, reduction of elevated cancer risk in selected patients, and treatment of disease. This page provides general educational information; the appropriate procedure, eligibility and timing are determined through individual medical assessment.

When the goal is chest contouring rather than treatment of disease, the technique is tailored to the amount of breast tissue, skin laxity and the desired chest shape. Approaches may include a double-incision technique with nipple grafting, or a periareolar (keyhole) approach in suitable anatomy. The surgeon also considers scar placement, chest-wall contour and future revision options.

What to discuss

Questions for your consultation

Which technique suits you

The approach depends on tissue volume, skin elasticity and the contour you want.

Scars and nipple position

Where incisions fall and how nipple–areola position and sensation are planned.

Risks

Bleeding, infection, contour irregularity, sensation changes and the possibility of revision.

Recovery

Drain use, compression garments, activity restrictions and follow-up schedule.

Eligibility assessment

Medical and psychological assessment where required, and any eligibility criteria that apply.

Future procedures

Whether staged or additional surgery may be needed to refine the result.

Assessment comes first

Chest surgery is elective and requires assessment by an appropriately qualified surgeon, including a discussion of risks, alternatives, realistic outcomes and — where required — eligibility criteria. No outcome can be guaranteed.

Questions

Mastectomy & chest surgery FAQs

What is the difference between mastectomy and breast reduction?

Breast reduction removes a portion of tissue to make the breasts smaller while preserving a feminine breast shape. Mastectomy for chest contouring removes breast tissue to create a flatter, more masculine chest wall, with a different approach to nipple position and scars.

Will I need follow-up surgery?

Most people are satisfied with a single procedure, but refinement or revision surgery is sometimes desired for contour or scar reasons. This is discussed in advance.

How long is recovery?

Recovery varies with the technique and individual healing. Compression garments, limited arm activity and drain care are commonly required for a period after surgery, with a gradual return to full activity as advised by your surgeon.

Your journey starts with information

Arrange a confidential consultation to discuss chest surgery

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

Breast Augmentation: Implants & Fat Transfer | Breast Enlargement Guide

Page: /surgical-breast-enhancement/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Surgical treatment information

Surgical Breast Enhancement: Implants & Fat Transfer

Surgical options can increase breast volume or alter breast shape. Like all surgery, they carry risks, require informed consent and may involve future procedures.

Breast augmentation is a surgical procedure designed to increase breast volume or alter breast shape using implants or, in selected cases, the patient's own fat. It may be considered by people seeking greater fullness, a change in proportions, or restoration of volume following pregnancy or weight change.

Treatment planning should take account of anatomy, existing breast tissue, skin characteristics, general health, lifestyle, expectations, and the individual's willingness to accept surgical risks and future follow-up. An appropriately credentialed surgeon should lead this assessment.

Options

Surgical pathways

[Image: Abstract breast-focused medical artwork in burgundy, rose and champagne gold]

Breast augmentation

A personalised approach to increasing breast volume or altering shape using implants or, in selected cases, your own fat.

[Image: Abstract breast contour and implant-inspired medical artwork in burgundy and rose]

Breast implants

Medical devices with an outer shell and a silicone gel filling — not the same as injectable silicone.

[Image: Stylised breast tissue illustration with bold gold and rose details]

Fat transfer

Using your own fat, transferred from a donor area, for a modest and individualised increase in volume.

More procedures

Revision, reduction, lift and corrective surgery

[Image: Abstract breast anatomy contours in rose against a dark burgundy ground]

Breast revision

Implant exchange, removal or repair when a previous operation needs attention.

[Image: Abstract breast anatomy contours in rose against a dark burgundy ground]

Breast reduction

Reducing breast size and weight to relieve physical symptoms and restore proportion.

[Image: Abstract breast contour illustration with rose and champagne-gold lift arcs]

Breast lift (mastopexy)

Reshaping and raising sagging breasts without adding volume.

[Image: Abstract breast anatomy contours in rose against a dark burgundy ground]

Mastectomy & chest surgery

Removal of breast tissue for chest contouring, risk reduction or treatment.

[Image: Abstract breast-medical illustration with rose and gold contour lines]

Breast filler removal

Corrective surgery for injected fillers and liquid silicone that have caused problems.

What patients should understand

Key considerations before surgery

Implant vs. fat transfer

The difference between implant-based augmentation and autologous fat transfer.

Qualified surgeon

The importance of a qualified and appropriately credentialed plastic surgeon.

Preoperative assessment

The need for preoperative assessment and informed consent.

Recovery & scarring

Recovery requirements, scar management and follow-up.

Complications & revision

Potential complications and the possibility of revision surgery.

No guarantees

Surgery cannot promise permanent outcomes, guaranteed symmetry or zero scarring.

Preparation

Preparing for a surgical consultation

Come prepared to discuss your goals, health and the questions that matter most to you.

[Image: true]

Clarify your goals and expectations

What change are you hoping for, and how would you prioritise proportion, size and shape?

Share your full medical history

Including previous surgery, medications, allergies and relevant family history.

Ask about credentials

Request the surgeon’s qualifications, registration and specific breast-procedure experience.

Ask about the plan and its limits

Discuss technique, expected limitations, recovery, costs and potential revision.

Take time to decide

Proceed only when you understand the risks, alternatives and aftercare, and feel ready to consent.

No guarantees

Surgery is not appropriate for everyone and cannot promise permanent outcomes, guaranteed symmetry, zero scarring or a risk-free result. Individual suitability, risks and outcomes vary and require assessment by an appropriately qualified clinician.

Your journey starts with information

Discuss surgical options with a qualified surgeon

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss them with a qualified, registered healthcare professional who can assess you in person.

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Page: /terms-of-use/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

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Understanding Breast Enlargement: An Evidence-Aware Overview

Page: /understanding-breast-enlargement/

[Image: Bold abstract breast-medical banner artwork with rose anatomy contours and champagne-gold highlights]

Start here

Understanding Breast Enlargement

Before comparing treatments, it helps to understand what breast enlargement actually involves — and the difference between changing how the breasts look and changing the breast itself.

“Breast enlargement” describes two very different things. One is a change in the look of the breasts — a bra, padding, posture or clothing. The other is a change in the breast itself — more glandular or fatty tissue, or a device placed inside it. Confusing the two is the single most common reason people spend money on something that cannot work.

This page sets out the vocabulary, the biology and the framework used throughout the rest of the site, so that every treatment page and evidence verdict has a clear reference point.

The core distinction

Appearance vs. tissue

Appearance only

A correctly fitted bra, padding, push-up styles, inserts, styling and posture change how the chest looks immediately. They carry no medical risk, are fully reversible, and do not alter breast tissue.

Real tissue or a device

Fat transfer moves your own fat into the breast; implants place a prosthesis. Both are procedures with recovery, risk and cost. Only these routes add lasting volume without hormones.

What the breast is made of

The breast sits on the chest wall over the pectoral muscles. It contains glandular tissue (lobules and ducts that produce and carry milk), fibrous connective tissue that supports it, and fat. There is no muscle inside the breast — a point that explains why chest exercises firm the wall but do not enlarge the breast. Skin and suspensory ligaments give the breast its shape and are the structures that stretch with pregnancy, weight change and age.

Breast size is therefore determined by how much glandular and fatty tissue sits on the chest, plus the skin envelope around it. Nothing applied to the surface of the skin can grow the tissue underneath.

How breast tissue actually grows

Breast development is driven mainly by hormones — above all oestrogen — during puberty, with further changes in pregnancy and breastfeeding. In adult life, breast volume mostly follows overall body fat, the hormonal cycle, pregnancy and age. The signal that builds breast tissue is internal and hormonal.

This is why the only known routes to more tissue are hormonal stimulation (a medical matter, not a cosmetic one), fat transfer (your own fat, moved by a surgeon), and implants (a prosthesis). Creams, pills, massage, suction cups and devices have no established mechanism for adding breast tissue at all.

A quick test: swelling or growth?

- If fullness fades within hours or days, it was fluid, swelling or a cycle effect — not growth.
- If it appears and disappears with your menstrual cycle, it is a normal cyclical change.
- If it follows weight gain, the fat changed — and that would happen without any product.
- Lasting added volume appears only after a procedure that places tissue or material in the breast, or after hormonal development.

At a glance

The four broad routes to more volume

Route What it is Adds lasting tissue? Nature Cosmetic Bras, padding, styling, posture No — changes appearance only Free or low cost, reversible, no medical risk Fat transfer Your own fat moved by liposuction and injection Yes — modest and variable Minimally invasive procedure with real risks Implants A saline or silicone device placed surgically Yes — predictable volume Surgery with long-term monitoring and revision risk Hormonal Medically supervised hormone treatment Yes, in principle A medical treatment, not a cosmetic route; carries systemic risks
Non-surgical injectables such as fillers, PRP and biostimulators do not belong on this list as reliable routes to lasting volume — see the evidence guide for why.

How to use this website

A clear path from curiosity to an informed decision

Work through the information at your own pace. No decision about your body should ever be rushed by advertising.

[Image: Abstract breast-medical artwork with rose tissue forms and gold contour lines]

Understand the biology

Read this page and the breast anatomy guide so that you can tell appearance changes from tissue changes.

Learn each option

Work through the non-surgical and surgical sections to understand intended effects, mechanisms and limitations.

Check the evidence

Use the evidence guide and research libraries to see what is proven, what is unproven and what to avoid.

Understand the risks

Review candidacy, contraindications, complications and the warning signs that need medical attention.

Prepare for a clinician

Take written questions to a registered, appropriately qualified professional and compare more than one opinion.

Educational information only — not medical advice

This page explains concepts for general education. It cannot account for your health, anatomy or circumstances, and it does not recommend any treatment. Decisions about breast enlargement should be made with a qualified, registered clinician after proper assessment.

Your journey starts with information

Ready to look at the options in detail?

Use the guides, comparisons and research libraries on this website to prepare your questions — then discuss

them with a qualified, registered healthcare professional who can assess you in person.

Research dataset: asps-breast-enlargement

American Society of Plastic Surgeons (ASPS) — https://www.plasticsurgery.org/

generated: 2026-10-20

source: American Society of Plastic Surgeons (ASPS)

sourceUrl: https://www.plasticsurgery.org/

about: American Society of Plastic Surgeons patient information related to breast enlargement: breast augmentation (augmentation mammaplasty), candidacy, consultation, questions to ask, cost, breast implant types, risks and safety, preparation, procedure steps, recovery, results, choosing a plastic surgeon and fat transfer breast augmentation.

categories

Record 1

id: augmentation

name: Breast augmentation

description: The ASPS overview of breast augmentation: what the operation is, what it can and cannot do, who makes a suitable candidate, what happens at consultation and the questions patients are advised to ask.

Record 2

id: implants

name: Breast implants

description: ASPS information on the implants themselves: fill types (saline, silicone, structured), shapes and surfaces (form-stable, round, smooth, textured), FDA age approvals and surface-related safety concerns.

Record 3

id: safety

name: Risks and safety

description: ASPS information on the risks of augmentation and implant-specific safety concerns, including capsular contracture, rupture, BIA-ALCL, BIA-SCC, breast implant illness and FDA screening intervals.

Record 4

id: surgery-and-recovery

name: Preparation, procedure and recovery

description: Practical steps around the operation itself: pre-operative preparation, the five surgical steps (anaesthesia, incision, placement, closure, result), immediate aftercare and the recovery timeline.

Record 5

id: fat-transfer

name: Fat transfer breast augmentation

description: ASPS information on enlarging the breasts with your own fat: liposuction harvesting, reinjection, who it suits and the research it references.

Record 6

id: cost-and-surgeon

name: Cost and choosing a surgeon

description: What ASPS publishes on augmentation fees, what is and is not included, insurance coverage, and the credentials an ASPS member plastic surgeon must hold.

documents

Record 1

id: breast-augmentation

title: Breast Augmentation (Augmentation Mammaplasty)

category: augmentation

kind: Procedure overview

url: https://www.plasticsurgery.org/cosmetic-procedures/breast-augmentation

summary: The ASPS overview of breast augmentation: a procedure that uses breast implants or fat transfer to increase breast size. It explains that augmentation can restore volume lost after weight reduction or pregnancy, create a rounder shape or correct natural asymmetry, lists what the surgery can and cannot achieve, and points patients to implant safety, fat transfer and breast lift information.

keyPoints

- Breast augmentation (also called augmentation mammaplasty) uses breast implants or fat transfer to increase the size of the breasts.
- It can restore volume lost after weight reduction or pregnancy, achieve a rounder shape, or improve natural size asymmetry.
- When fat from another part of the body is used, the procedure is called fat transfer breast augmentation.
- Surgery can increase fullness and projection, improve the balance of breast and hip contours, and enhance self-image and confidence.
- Augmentation does not correct severely drooping breasts; a breast lift may be required alongside it for sagging breasts to look fuller and lifted.
- A lift can often be done at the same time as augmentation, or may require a separate operation.
- Implants are also used for breast reconstruction after mastectomy or injury.

Record 2

id: candidates

title: Breast Augmentation Candidates

category: augmentation

kind: Patient guidance

url: https://www.plasticsurgery.org/cosmetic-procedures/breast-augmentation/candidates

summary: ASPS guidance on who may be a candidate for breast augmentation. The page stresses that the procedure is deeply personal and should be chosen for oneself, lists the medical and developmental factors surgeons look for, and describes the common reasons people seek augmentation.

keyPoints

- The decision is deeply personal and should be made for yourself rather than to satisfy someone else.
- Candidates should be physically healthy and not pregnant or breastfeeding.
- Breasts should be fully developed and expectations should be realistic.
- Common motivations include dissatisfaction with small size, loss of shape or volume after pregnancy or weight loss, and changes with aging.
- Other reasons include an empty upper breast, asymmetry, or one or both breasts failing to develop normally.

- Reviewing before-and-after photos and recovery information beforehand helps set reasonable expectations.

Record 3

id: consultation

title: Breast Augmentation Consultation

category: augmentation

kind: Patient guidance

url: https://www.plasticsurgery.org/cosmetic-procedures/breast-augmentation/consultation

summary: What to expect at a breast augmentation consultation: the topics patients should be ready to discuss, the medical history the surgeon collects, and the physical assessment and measurements used to make a recommendation.

keyPoints

- Be ready to discuss your motivation, expectations and the outcome you want.
- Disclose medical conditions, allergies, previous treatments, medications, supplements and any substance use.
- Share family history of breast cancer and the results of previous mammograms or biopsies.
- The surgeon evaluates general health and risk factors, then takes detailed breast measurements and photographs.
- The examination covers size, shape, skin quality and nipple-areola placement before treatment is recommended.
- The surgeon discusses the expected outcome, risks and complications, and offers a written question checklist.

Record 4

id: questions

title: Questions to Ask Your Plastic Surgeon

category: augmentation

kind: Question checklist

url: https://www.plasticsurgery.org/cosmetic-procedures/breast-augmentation/questions

summary: The ASPS checklist of questions patients should take to a breast augmentation consultation, covering surgeon credentials, facility accreditation, the surgical plan, recovery support, risk handling and long-term considerations.

keyPoints

- Verify board certification by the American Board of Plastic Surgery and plastic-surgery-specific training.
- Ask about years of surgical training and where the surgeon has hospital privileges.
- Confirm the surgical facility is accredited, state-licensed or Medicare-certified.
- Discuss candidacy, the recommended implant shape, size and texture, and incision and placement choices.
- Ask how long recovery takes, what help you will need, the risks, and how complications would be handled.
- Ask about possible future operations, effects on breastfeeding, and how results change over time and after pregnancy.

Record 5

id: cost

title: Breast Augmentation Cost

category: cost-and-surgeon

kind: Cost guidance

url: https://www.plasticsurgery.org/cosmetic-procedures/breast-augmentation/cost

summary: ASPS guidance on the cost of breast augmentation, including its published average surgeon fees for implant and fat-grafting augmentation, the expenses those figures exclude, and the insurance position for cosmetic surgery.

keyPoints

- Published average costs reflect surgeon fees only, not the total price of the procedure.
- Anesthesia, operating-room or facility fees, medical tests, garments, prescriptions and related expenses are excluded.
- The surgeon fee varies with experience, procedure type and geographic location.
- Many surgeons offer patient financing plans.
- Most insurance plans do not cover cosmetic augmentation, related complications or revision surgery.
- Some carriers exclude breast-disease coverage for implant patients.

Record 6

id: implants

title: Types of Breast Implants

category: implants

kind: Device overview

url: https://www.plasticsurgery.org/cosmetic-procedures/breast-augmentation/implants

summary: ASPS guidance on the main implant options: fill type (saline, silicone gel, structured saline) and shape or surface (form-stable "gummy bear", round, smooth, textured), with the FDA age approvals and the main trade-offs of each choice.

keyPoints

- Saline implants are filled with sterile salt water; if they leak the implant collapses and the body safely absorbs the fluid.
- Silicone gel implants are designed to mimic the feel of natural tissue; a leak may stay contained and the implant does not collapse.
- Structured saline implants contain an inner matrix intended to give a more natural feel.
- Form-stable "gummy bear" implants keep their shape even if the shell breaks; they use a firmer gel, need a longer incision and shaped versions can rotate.
- Round implants look fuller and do not rotate out of place; smooth implants feel softest but may ripple, while textured implants grip tissue.
- Textured surfaces are the surface most associated with breast implant-associated anaplastic large cell lymphoma (BIA-ALCL).

Record 7

id: safety

title: Breast Augmentation Risks and Safety

category: safety

kind: Safety guidance

url: https://www.plasticsurgery.org/cosmetic-procedures/breast-augmentation/safety

summary: The full list of risks ASPS publishes for breast augmentation — from anesthesia complications and capsular contracture to implant-associated cancers and breast implant illness — together with the reminder that implants are not lifetime devices and that FDA-recommended screening applies to silicone implants.

keyPoints

- Common risks include anesthesia complications, bleeding or hematoma, infection, capsular contracture, implant rupture, changes in sensation, poor scarring and possible revision surgery.
- Rare cancers of the implant capsule (BIA-ALCL and BIA-SCC) and systemic symptoms described as breast implant illness are acknowledged.
- Most people with implants never experience serious complications, but FDA-approved devices still carry documented risks.
- Implants are not guaranteed for life; future replacement surgery may be needed.
- Ongoing breast-health exams and implant-condition checks are needed, and pregnancy, weight loss and menopause change appearance over time.
- Routine screening matters because some ruptures produce no symptoms.

Record 8

id: preparation

title: Breast Augmentation Preparation

category: surgery-and-recovery

kind: Pre-operative guidance

url: https://www.plasticsurgery.org/cosmetic-procedures/breast-augmentation/preparation

summary: The medical and lifestyle steps ASPS advises before breast augmentation: tests and medication changes, stopping substances that increase bleeding, choosing an accredited surgical facility and arranging post-operative support.

keyPoints

- You may need blood tests and adjustments to your medications, including stopping certain drugs.
- Stop smoking and avoid aspirin and anti-inflammatory drugs that can increase bleeding.
- Recreational drugs such as cocaine must be stopped before surgery.
- Breast augmentation should be performed in an accredited outpatient or ambulatory surgical center or a hospital.
- If going home the same day, arrange a driver and someone to stay with you for at least the first night.

Record 9

id: procedure

title: Breast Augmentation Procedure Steps

category: surgery-and-recovery

kind: Procedure steps

url: https://www.plasticsurgery.org/cosmetic-procedures/breast-augmentation/procedure

summary: The five steps of breast augmentation as set out by ASPS — anaesthesia, incision, implant placement, closing the incisions and the immediate result — with the incision and placement options and a caution about the transumbilical approach.

keyPoints

- Step 1: anesthesia with IV sedation or general anesthesia, selected for the patient.
- Step 2: incisions placed in discreet locations — around the areola (peri-areolar), in the breast fold (inframammary) or in the armpit (axillary).
- Step 3: the implant is placed either under the pectoral muscle (submuscular) or behind the breast tissue over the muscle (submammary).

- Step 4: incisions are closed with layered sutures plus skin sutures, adhesive or tape; scar quality depends on genetics, nicotine use and infection.
- Step 5: results are visible immediately after surgery.
- The belly-button (transumbilical) incision approach is associated with a higher complication rate.

Record 10

id: recovery

title: Breast Augmentation Recovery

category: surgery-and-recovery

kind: Recovery guidance

url: https://www.plasticsurgery.org/cosmetic-procedures/breast-augmentation/recovery

summary: What to expect straight after breast augmentation: dressings and support garments, monitoring and discharge timing, the instructions and medications you go home with, and the pain and activity timeline during healing.

keyPoints

- Breasts are wrapped in gauze with an elastic bandage or support bra to limit swelling.
- Patients are monitored in recovery and typically discharged about an hour after surgery once stable.
- You receive written post-operative instructions, a follow-up appointment and possibly pain medication and an antibiotic.
- The support garment or bandeau should be worn around the clock as directed; incision-care instructions vary by surgeon.
- Take it easy for the first several days because of bleeding risk around the implant pockets, and resume exercise only when cleared.
- Acute pain usually subsides after one to five days, while soreness and swelling may persist for a few weeks.

Record 11

id: results

title: Breast Augmentation Results

category: surgery-and-recovery

kind: Results guidance

url: https://www.plasticsurgery.org/cosmetic-procedures/breast-augmentation/results

summary: How breast augmentation results settle over time: shape finalising over the first weeks as swelling resolves, incision marks fading slowly, the follow-up cadence, and the fact that implants are not lifetime devices and results change with aging.

keyPoints

- Breasts are larger right away, but the final shape takes a few weeks as swelling subsides and the skin stretches.
- Some patients, such as those with asymmetry or very small starting breasts, may need a bandeau to help shape the result.
- Incision lines can take months — even a couple of years — to fade.
- Optimal results depend on following post-operative instructions and attending follow-up visits.
- Implants are not lifetime devices; annual exams evaluate breast health and implant integrity.
- Aging, weight fluctuation, hormones and gravity change the result; some patients later choose a lift or implant exchange.

Record 12

id: surgeon

title: Choosing a Plastic Surgeon

category: cost-and-surgeon

kind: Credential guidance

url: https://www.plasticsurgery.org/cosmetic-procedures/breast-augmentation/surgeon

summary: The credentials ASPS says patients should look for: an ASPS member plastic surgeon board-certified by the American Board of Plastic Surgery (or the Royal College of Physicians and Surgeons of Canada), with the required training, exams, continuing education and accredited facility standards.

keyPoints

- Choose an ASPS member plastic surgeon board-certified by the American Board of Plastic Surgery or the Royal College of Physicians and Surgeons of Canada.
- Certification requires at least six years of surgical training after medical school, including a minimum of three years of plastic surgery residency.
- Surgeons must pass comprehensive oral and written exams and graduate from an accredited medical school.
- Continuing medical education including patient safety must be completed every year.
- Surgery must be performed in accredited, state-licensed or Medicare-certified facilities.
- The ABPS is recognized by the ABMS, and no ABMS-recognized certifying board has "cosmetic surgery" in its name.

Record 13

id: fat-transfer

title: Fat Transfer Breast Augmentation

category: fat-transfer

kind: Procedure overview

url: https://www.plasticsurgery.org/cosmetic-procedures/fat-transfer-breast-augmentation

summary: The ASPS page on fat transfer breast augmentation: liposuction is used to harvest fat from elsewhere in the body and reinject it into the breasts, offered as an option for people wanting a relatively small, natural-looking increase in size.

keyPoints

- Fat is taken by liposuction from other parts of the body and injected into the breasts to enhance them.
- It is an option for people looking for a relatively small increase in breast size.
- It is described as a choice for those who would prefer natural results.
- ASPS cites research on pre-expansion for mega-volume transfer, fat transfer as an alternative to implants, and techniques that substantially increase breast size using the patient's own fat.
- When fat transfer is used, the procedure is referred to as fat transfer breast augmentation rather than augmentation mammaplasty with implants.

facts

Record 1

topic: Average cost with implants

statement: ASPS reports an average surgeon fee of $4,875 for breast augmentation with implants.

source: cost

Record 2

topic: Average cost with fat grafting

statement: ASPS reports an average surgeon fee of $5,719 for augmentation by fat grafting.

source: cost

Record 3

topic: What the average covers

statement: Quoted averages are surgeon fees only; anesthesia, facility fees, tests, garments and prescriptions are excluded.

source: cost

Record 4

topic: Insurance cover

statement: Most insurance plans do not cover cosmetic augmentation, its complications or revision surgery.

source: cost

Record 5

topic: Saline age approval

statement: The FDA approves saline implants for augmentation in women aged 18 or older.

source: implants

Record 6

topic: Silicone age approval

statement: The FDA approves silicone implants for augmentation in women aged 22 or older.

source: implants

Record 7

topic: Textured surface and BIA-ALCL

statement: Breast implant-associated anaplastic large cell lymphoma occurs most frequently with textured-surface implants.

source: safety

Record 8

topic: First silicone screening

statement: The FDA recommends MRI or ultrasound screening 5-6 years after silicone implant placement.

source: safety

Record 9

topic: Ongoing silicone screening

statement: After the first screen, the FDA recommends screening every 2-3 years.

source: safety

Record 10

topic: Silent rupture

statement: Some women with a ruptured implant have no symptoms, which is why routine screening is advised.

source: safety

Record 11

topic: Implant longevity

statement: Implants are not guaranteed for life and future replacement surgery may be needed.

source: safety

Record 12

topic: Facility standard

statement: Augmentation should be performed in an accredited outpatient or ambulatory surgical center or a hospital.

source: preparation

Record 13

topic: Substances and bleeding

statement: Smoking, aspirin, anti-inflammatory drugs and recreational drugs such as cocaine should be stopped before surgery because they increase bleeding.

source: preparation

Record 14

topic: Anesthesia

statement: The procedure is performed under IV sedation or general anesthesia.

source: procedure

Record 15

topic: Incision options

statement: Incisions may be peri-areolar, in the inframammary fold or in the armpit (axillary).

source: procedure

Record 16

topic: Placement options

statement: Implants are placed either under the pectoral muscle or behind the breast tissue over the muscle.

source: procedure

Record 17

topic: Transumbilical approach

statement: The belly-button incision approach is associated with a higher complication rate.

source: procedure

Record 18

topic: Discharge timing

statement: Patients typically go home about an hour after surgery once they are stable.

source: recovery

Record 19

topic: Acute pain

statement: Acute pain usually subsides after 1-5 days.

source: recovery

Record 20

topic: Soreness and swelling

statement: Soreness and swelling may persist for a few weeks after surgery.

source: recovery

Record 21

topic: Final result timeline

statement: Final results may take a few weeks to appear as swelling subsides and the skin stretches.

source: results

Record 22

topic: Scar maturation

statement: Incision lines can take several months, up to a couple of years, to fade.

source: results

Record 23

topic: Follow-up

statement: Annual examinations are suggested to assess breast health and implant integrity.

source: results

Record 24

topic: Surgeon training

statement: ASPS member surgeons complete at least six years of surgical training after medical school, including at least three years of plastic surgery residency.

source: surgeon

Record 25

topic: Board certification

statement: Patients are advised to verify certification by the American Board of Plastic Surgery, which is recognized by the ABMS.

source: surgeon

Record 26

topic: Facility credentials

statement: ASPS member surgeons operate in accredited, state-licensed or Medicare-certified surgical facilities.

source: surgeon

Record 27

topic: Fat transfer method

statement: Fat transfer augmentation uses liposuction to take fat from elsewhere in the body and inject it into the breasts.

source: fat-transfer

Record 28

topic: Fat transfer suitability

statement: Fat transfer is presented as an option for a relatively small increase in size for those wanting natural results.

source: fat-transfer

Research dataset: baaps-breast-enlargement

British Association of Aesthetic Plastic Surgeons (BAAPS) — https://baaps.org.uk/

generated: 2026-10-20

source: British Association of Aesthetic Plastic Surgeons (BAAPS)

sourceUrl: https://baaps.org.uk/

about: BAAPS patient information related to breast enlargement (breast augmentation): the Your Guide to Breast Augmentation booklet produced jointly with ABS and BAPRAS, the BAAPS breast augmentation procedure page, breast implant safety (BII, BIA-ALCL, BIA-SCC, PIP implants) and BAAPS consumer safety guidelines.

categories

Record 1

id: augmentation

name: Breast augmentation

description: BAAPS patient information on breast enlargement itself: what the operation involves, consultation and consent, techniques, implant choices, recovery and expected outcomes.

Record 2

id: booklet

name: Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS)

description: The joint patient booklet from the Association of Breast Surgery, BAAPS and BAPRAS (Edition 2, 2021): a section-by-section guide covering decisions, surgery, implants, complications, long-term consequences, alternatives and the national implant registry.

Record 3

id: safety

name: Breast implant safety

description: BAAPS information on breast implant associated conditions — Breast Implant Illness (BII), BIA-ALCL, BIA-SCC, PIP implants — and the association's published safety statements.

Record 4

id: consumer-safety

name: Consumer safety guidelines

description: BAAPS fourteen-point consumer safety guidance for anyone considering aesthetic surgery, from making your own decision to checking credentials and taking your time.

documents

Record 1

id: breast-augmentation

title: Breast Augmentation

category: augmentation

kind: Procedure page

url: https://baaps.org.uk/patients/procedures/1/breast_augmentation

updated: 2021

summary: The BAAPS procedure page for breast augmentation (bilateral breast augmentation, BBA) — an operation to enlarge the breasts, usually by inserting an implant beneath the breast tissue. The page describes the procedure as potentially life-enhancing but stresses that it is a big decision, major surgery with potential risks, and that the hoped-for result cannot be guaranteed. It links to the full joint patient booklet and lists the common names patients use for the procedure.

keyPoints

- Breast augmentation (BBA) is an operation to enlarge the breasts, usually by inserting an implant beneath the breast tissue.
- It can be a life-enhancing procedure, but having a breast enlargement is a big decision.
- It is major surgery with potential risks, and the result you may hope for cannot be guaranteed.
- The joint ABS, BAAPS and BAPRAS booklet explains the procedure and the short- and long-term risks; any further questions after consultation should be asked of your surgeon.
- The procedure is also commonly known as a boob job, enlargement, implant or breast size.

Record 2

id: guide-booklet

title: Your Guide to Breast Augmentation (ABS, BAAPS & BAPRAS, Edition 2, 2021)

category: booklet

kind: Patient information booklet

url: https://baaps.org.uk/_userfiles/pages/files/print_bapras_breast_aug_2021_final_revised_3.pdf

updated: 2021-04

summary: The joint patient booklet from the Association of Breast Surgery, BAAPS and BAPRAS. It covers who the guide is for and what breast augmentation involves; how to seek a consultation, check a surgeon's qualifications, understand costs and the cooling-off period; the surgery itself including implant placement (sub-glandular, sub-muscular, dual plane), operation time and recovery; implant type, size, shape and texture; a full list of complications with typical frequencies; long-term consequences including mammography, BIA-ALCL and breast implant illness; alternatives such as lipofilling and mastopexy; and the UK Breast and Cosmetic Implant Registry. Editors: Lee Martin and Emma de Sousa; reviewed by PRASEAG.

keyPoints

- Breast augmentation is usually done by inserting an implant beneath the breast to make it look larger; it can enlarge always-small breasts or fill out breasts that have emptied, for example after pregnancy.
- It will reshape breasts to some extent and can also be used to correct unequal breast size.
- Fat transfer (lipomodelling, lipofilling or fat grafting) is an alternative in a few people but is not suitable for all and often requires more than one episode of surgery.
- Patients tend to be pleased with results, but you need realistic expectations and must understand what can and cannot be achieved, the limitations of surgery and the long-term consequences of implants.
- Your initial consultation should be with the surgeon who will do the operation; your surgeon will normally seek permission to write to your GP; a consultation fee is normal.
- At consultation you will be examined, measurements taken, and implant shapes, sizes and pocket positions discussed; some surgeons use implant sizers in a bra or 3D photography, which is only a guide — the same implant can look very different in different people.
- You should be given a cooling-off period of at least two weeks before surgery and offered a second consultation; you should not feel rushed.
- Breast augmentation is a lifetime commitment, not a one-stop permanent solution: you will need revision surgery

as the years go by and must be prepared personally and financially.

- Nobody needs an urgent breast augmentation — if you are not offered a cooling-off period or are put under pressure, walk away.
- Surgeons should be listed on the GMC specialist register (gmc-uk.org); membership of ABS, BAAPS and/or BAPRAS indicates a standard set of recognised qualifications.
- Internet search listings may be promotional; a prominent listing measures marketing ability, not service quality.
- Cosmetic breast augmentation is not available on the NHS except in exceptional circumstances such as very uneven breasts, no breasts, or as part of reconstruction; after surgery abroad the NHS will only deal with emergency complications on return.
- Breast enlargement involves placing an implant under the breast tissue; implants are usually inserted through an incision under the breast at the crease, but can also go in via the armpit or around the nipple.
- Implants can sit behind the breast on top of the chest wall muscle (sub-glandular) or behind the breast and muscle (sub-muscular); your surgeon will advise which is appropriate.
- Sub-glandular placement is considered the simplest enlargement procedure and less likely to cause significant discomfort; it is also effective with some drooping.
- Sub-muscular placement provides more padding for the implant, helpful for slender patients and those with little breast tissue, reducing the chances of feeling or seeing the implant.
- Dual plane augmentation combines the two routes, placing implants partly behind the breast and partly behind the muscle; implants behind the muscle may move with muscle contraction (animation), which is normal.
- Breast enlargement surgery takes about one-and-a-half hours, usually under general anaesthetic; you may go home the same day but many patients spend one night in hospital.
- You will be mobile from day one and should be back to full exercise within six weeks; one to two weeks off work is recommended.
- At first the breasts may look high and the skin tight; this tends to settle over the first six weeks, with the long-term result emerging over 3-4 months as a more natural shape appears.
- Scars tend to be red in the first six weeks, change to purple over the next three months, then fade to white; abnormal scarring can occur.
- All implant shells are made of silicone; some have an additional polyurethane coating; shells are filled with silicone gel or saline, and implants have been used since the 1960s with evidence suggesting they are safe.
- Most patients are offered silicone gel-filled implants — they feel most natural, have a range of shapes and are durable; saline implants tend to feel less natural, ripples may be more visible and they carry a risk of deflation; all other filler materials have been withdrawn for use in the UK.
- Implants are supplied by volume and it is not possible to guarantee a cup size; the larger the implant and the slimmer the patient, the less natural the result, and an out-of-proportion implant is more likely to cause drooping, tissue stretching and palpability.
- Implants are round or teardrop-shaped (anatomical); round implants give bigger volume at the top and are the same width as tall, while teardrop implants allow width and height to be chosen separately; both come in varying degrees of projection.
- Texturing reduces capsular contracture (particularly with implants placed in front of the muscle) and reduces movement or rotation; anatomical implants tend to be textured because rotation would alter the shape, while rotation of a round implant is not noticeable.
- Textured surfaces are related to BIA-ALCL, an uncommon condition; smooth-surface implants currently appear to have no link to BIA-ALCL; smooth implants are almost always round and can take several months to settle, and may slip sideways (lateral displacement) or drop below the crease (bottoming out).
- Small quantities of silicone from implants can be taken up by the body and found in breast tissue and sometimes armpit lymph glands; silicone granuloma is inflammation and extra tissue around silicone deposits.
- Over time the implant shell may fail and gel may leak; many ruptures are discovered by accident, and a ruptured implant should be removed — replacement at the same or a later operation is a choice.
- There is no association between breast augmentation and breast cancer.

- Serious complications are uncommon, occurring in less than 1 in 100 operations.
- Bleeding (haematoma) occurs in less than 1 in 100, usually immediately but occasionally up to two weeks later; the breast becomes very swollen and tight and you return to theatre to have the blood removed — haematoma can increase the risk of later capsular contracture.
- Infection occurs in less than 1 in 1000 cases, usually apparent over the first two or three weeks; most patients with an established infection around the implant need the implant removed, and a new implant cannot be inserted immediately — three to six months should wait for the infection to resolve.
- Perfect symmetry is not an achievable goal; asymmetry can be of breast or nipple, size, shape and position, may appear more pronounced after surgery, and implants can settle differently in their pockets.
- Some patients have a naturally wide cleavage that implants will not improve; with sub-muscular placement the cleavage can appear wider, and breasts and implants may fall to the sides when lying down, which is normal.
- Stretch marks may develop, especially with larger implants, and veins may become more noticeable.
- Breasts will sag with time because they are larger and heavier; the larger the implant, the more the breast will droop, and breasts may bottom out.
- Implants do not interfere with the ability to breastfeed, though some evidence suggests milk production may be reduced in some women; there is no evidence of increased illness in children of women with silicone gel implants.
- Every patient forms a scar capsule around the implant; capsular contracture — when this scar contracts and makes the implant feel firmer — is up to 1 in 10 of all augmentations, but most of those patients will not need revision surgery.
- About 1% (1 in 100) a year need re-operation for any reason, so after 10 years about 10% will have had more surgery; capsular contracture is the most common reason and is likely to recur in 1 in 2 cases after secondary surgery.
- Around 1 in 5 patients will have reduced nipple sensation; recovery can take up to 12 months, but 1 in 10 will have some permanent numbness.
- It is common to be able to feel the implant, especially in slim patients or those with little breast tissue; this is an inevitable consequence of the operation and will not improve with time; lipomodelling can improve the situation in some patients.
- The American Core study (FDA Update 2011) suggested 1 in 10 implants had ruptured at 10 years, and half of all implants may rupture by 15 years; many ruptures are silent, contained within the capsule, and do not appear harmful.
- There is no universally agreed replacement schedule for implants and it is unusual to need exchange before ten years; if you have noticed no change you do not need regular follow-up or scans — but you should be prepared personally and financially for possible future surgery.
- Other reasons for re-operation include wanting larger implants, shape change with time or after pregnancy and weight fluctuation, rotation of teardrop implants, and intermittent swelling around implants.
- Long-term problems will not usually be covered by the package bought at the time of augmentation, and the NHS should not be expected to provide future private treatment.
- BIA-ALCL is recognised by the WHO as a condition associated with breast implants; it is not a breast cancer but a cancer of the scar tissue capsule, occurring between 2 and 28 years after insertion (average 8 years), usually presenting as swelling around the implant (seroma), and usually treated by removing the implant and capsule.
- Most BIA-ALCL cases worldwide have occurred with textured implants, with higher numbers where the texture is coarser; breast implants continue to have safety approval from the MHRA and FDA and continue to be used in reconstruction worldwide.
- Breast Implant Illness (BII) is a term used by some patients for symptoms including tiredness, brain fog, joint aches, immune-related symptoms, sleep disturbance, depression, hormonal issues, headaches, hair loss, chills, rash and neurological issues; it is not a medical diagnosis and there is no proven association with implants.
- Some patients report symptoms improve if implants are removed, but this is not true for all; research continues.
- Implants do not make breast self-examination more difficult, but they do interfere with mammography because X-rays cannot pass through the implant; tell the mammography service you have implants — special views or a

different centre may be used, and some patients opt for private MRI screening.

- It is safe to sunbathe and fly with implants; implant massage is not needed and is not recommended with modern textured implants; routine implant replacement after a specified time is not recommended.
- The only alternative surgical technique to enlarge the breast is lipofilling: fat removed by liposuction from hips, thighs or tummy and injected into the breast; only a relatively small amount can be injected so multiple operations are needed for a worthwhile effect, and some fat is absorbed in the initial weeks — fat that lasts brings permanent enlargement.
- Your own fat is the only substance that can be safely injected into the breast; other materials have been tried and withdrawn — do not allow anyone to inject anything other than your own fat.
- Breast uplift (mastopexy) is a different operation that lifts the nipple and breast tissue and tightens the skin without an implant; scarring is more obvious, and uplifted breasts tend to droop again with time and gravity.
- Mild droopiness can be improved by augmentation alone, but where volume loss and droopiness both bother you, augmentation alone will not solve drooping — your surgeon should discuss augmentation with mastopexy, as one operation or in two stages.
- In the UK the national Breast and Cosmetic Implant Registry records all implants placed or removed, allowing patients to be traced in the event of a future recall and identification of trends; entry requires consent and is strongly recommended.
- Avoid any deal with a non-refundable deposit or financial inducement to proceed — even refunding a consultation fee if you proceed with surgery is considered an inducement and against GMC guidance.
- A package price covering the entire process and the cost of treating complications in the initial weeks is appropriate; the cheapest deal may not be the best.
- Before your consultation it should be possible to obtain an approximate cost from your private hospital or the surgeon's secretary; after consultation(s) you should be given a written quote.

Record 3

id: implant-safety

title: Breast Implant Safety

category: safety

kind: Safety information page

url: https://baaps.org.uk/patients/safety_in_surgery/breast_implant_safety.aspx

summary: BAAPS information and guidance on breast implant associated conditions, with links to patient documents on Breast Implant Illness (BII) and BIA-ALCL, the association's updated BII statement of October 2022, joint statements on BIA-SCC and other breast implant associated lymphomas, PRASEAG guidance, the Allergan Natrelle textured implant withdrawal statement, the BAAPS/ABS/BAPRAS joint statement on breast implant safety, and BAAPS advice on PIP implants including signs and symptoms of rupture or inflammation.

keyPoints

- BAAPS publishes patient documents on Breast Implant Illness (BII) and Anaplastic Large Cell Lymphoma (BIA-ALCL), plus an updated BII statement from October 2022.
- In July 2023 BAAPS issued a joint statement on Breast Implant Associated Squamous Cell Carcinoma and other breast implant associated lymphomas.
- PRASEAG issued a statement on breast implants and guidance for patients following MHRA advice on BIA-ALCL, with BAAPS, BAPRAS and the Association of Breast Surgery working together.
- BAAPS issued statements on the withdrawal of Allergan (Natrelle) textured implants and a joint BAAPS/ABS/BAPRAS statement on breast implant safety (27 November 2018).
- On PIP implants the 2013 expert group found no evidence they were responsible for an increased rate of breast cancer; the gel was less cohesive than most contemporary implants and had a greater potential to cause local inflammation on rupture.
- Signs of PIP rupture, leak or inflammation include lumpiness of the breast, lumpiness or swelling of regional

lymph nodes, change in shape and size, redness, tenderness, swelling and firmness; symptoms may include pain and hypersensitivity.

- Patients unsure of their implant make should contact their surgeon or provider; patients with signs or symptoms of rupture or irritation should seek advice from their surgeon.
- BAAPS advised removal or replacement of PIP implants as a precaution because of the inferior quality of the gel, regardless of rupture rate.
- Breast implants continue to have safety approval from government organisations such as the UK MHRA and USA FDA and continue to be used in reconstruction worldwide.

Record 4

id: consumer-safety-guidelines

title: BAAPS Consumer Safety Guidelines

category: consumer-safety

kind: Safety guidance

url: https://baaps.org.uk/patients/safety_in_surgery/consumer_safety_guidelines.aspx

summary: BAAPS fourteen-point consumer safety guidance for anyone considering aesthetic surgery: make your own decision, do your research, ask the right questions at every stage, know your surgeon and check credentials on the GMC specialist register, beware pressurising selling and non-refundable deposits, expect a two-week cooling-off period, get a second opinion if needed, time the surgery well, think about location and travel, talk to your GP, remember you can change your mind, use common sense about special offers, and take your time.

keyPoints

- Only you can decide whether to go ahead with aesthetic surgery, but you need specialist input to understand what is possible, suitable and the risks and benefits.
- No procedure is 100% risk free and no surgeon can give a 100% guarantee of results; research the limitations and risks before any consultation.
- Know what will happen at every stage — pre-operative assessment, the day of surgery, aftercare, recovery time, follow-up and the possibility of further procedures.
- Your consultation should be with the actual surgeon who will operate; clinic managers and nurses should not be your only point of contact.
- A specialist plastic surgeon should have the FCRS (Plast) qualification and be listed on the Specialist Register of Plastic Surgeons with the GMC.
- Avoid any marketing or selling technique that financially or temporally traps you, including booking fees and non-refundable advances; your decision should be free of incentive.
- You should expect a two-week cooling-off period following your consultation to consider your options.
- Get a second opinion if necessary; a reputable surgeon will never apply a time limit, withhold key information or upsell additional surgery.
- Avoid surgery around major life events such as moving house, changing job, bereavement, relationship breakdown or the arrival of children.
- Give careful consideration to travelling long distances or overseas for surgery; travel insurance often will not cover treatment or extra nights for elective aesthetic procedures.
- Your GP can offer unbiased advice with no vested interest and may discuss functional, psychological and social benefits.
- You can cancel right up until the time you go to sleep for surgery.
- Don't be tempted by special offers or rushed into a quick surgery slot; anything that seems too good to be true probably is.
- Take as much time as you need after an initial consultation; a surgeon might recommend two consultations

before booking a procedure.

facts

Record 1

topic: What breast augmentation is

statement: Breast augmentation (bilateral breast augmentation or BBA) is an operation to enlarge the breasts, usually by inserting an implant beneath the breast tissue; it is also commonly known as a boob job, enlargement, implant or breast size.

source: breast-augmentation

Record 2

topic: Realistic expectations

statement: It can be a life-enhancing procedure, but having a breast enlargement is a big decision: it is major surgery with potential risks, and the result you may hope for cannot be guaranteed.

source: breast-augmentation

Record 3

topic: Who the booklet is for

statement: The joint ABS, BAAPS and BAPRAS booklet is for those looking for more information about breast augmentation, designed to be used alongside discussions with your surgeon, GP, family and friends.

source: guide-booklet

Record 4

topic: Indications

statement: Augmentation will enlarge breasts that have always been small and can fill out breasts that used to be larger, for example after pregnancy; it will reshape breasts to some extent and can correct unequal size.

source: guide-booklet

Record 5

topic: Consultation

statement: Your initial consultation should be with the surgeon who will do the operation; you will be examined and measurements taken, and implant shapes, sizes and pocket positions discussed; sizers or 3D photography are only a guide because the same implant can look very different in different people.

source: guide-booklet

Record 6

topic: Cooling-off period

statement: You should be given a cooling-off period of at least two weeks before surgery and offered a second consultation; nobody needs urgent breast augmentation, and if you are pressured to proceed you should walk away.

source: guide-booklet

Record 7

topic: Lifetime commitment

statement: Breast augmentation is a lifetime commitment and must not be considered a one-stop permanent solution; you will need revision surgery as the years go by and must be prepared personally and financially.

source: guide-booklet

Record 8

topic: Checking credentials

statement: All surgeons should be listed on the GMC specialist register with a licence to practise; surgeons may also be registered with ABS, BAAPS and/or BAPRAS, whose members hold a standard set of recognised qualifications.

source: guide-booklet

Record 9

topic: Cost and inducements

statement: You should avoid non-refundable deposits and financial inducements to proceed — even refunding a consultation fee if you book surgery is against GMC guidance; a written quote should follow consultation, and the cheapest deal may not be the best.

source: guide-booklet

Record 10

topic: NHS availability

statement: Cosmetic breast augmentation is not available on the NHS except in exceptional circumstances such as very uneven breasts, no breasts, or as part of reconstruction; after surgery abroad the NHS will only deal with emergency complications on return.

source: guide-booklet

Record 11

topic: Incisions

statement: Implants are usually inserted through an incision under the breast at the crease, but can also be put in via an incision in the armpit or around the nipple.

source: guide-booklet

Record 12

topic: Placement

statement: Implants sit either behind the breast on top of the chest wall muscle (sub-glandular) or behind the breast and muscle (sub-muscular); dual plane combines the two; animation of sub-muscular implants with muscle contraction is normal.

source: guide-booklet

Record 13

topic: Operation and recovery

statement: Surgery takes about one-and-a-half hours under general anaesthetic; many patients spend one night in hospital, are mobile from day one, back to full exercise within six weeks, and should take one to two weeks off work.

source: guide-booklet

Record 14

topic: Settling and scars

statement: Breasts may look high and tight at first, settling over six weeks with the long-term result emerging over 3-4 months; scars are red for six weeks, purple for three months, then fade to white.

source: guide-booklet

Record 15

topic: Implant materials

statement: All implant shells are silicone, filled with silicone gel or saline; most patients are offered silicone gel implants for natural feel and durability; all other filler materials have been withdrawn for use in the UK.

source: guide-booklet

Record 16

topic: Cup size

statement: Implants are supplied by volume and it is not possible to guarantee a cup size; larger implants on a slimmer frame look less natural and are more likely to cause complications such as drooping and tissue stretching.

source: guide-booklet

Record 17

topic: Shape and texture

statement: Implants are round or teardrop-shaped with varying projection; texturing reduces capsular contracture and rotation, anatomical implants tend to be textured, and textured surfaces are related to BIA-ALCL while smooth implants currently appear not to be.

source: guide-booklet

Record 18

topic: Serious complications

statement: Serious complications are uncommon with breast augmentation, occurring in less than 1 in 100 operations.

source: guide-booklet

Record 19

topic: Bleeding

statement: Bleeding (haematoma) risk is less than 1 in 100, usually immediate but occasionally up to two weeks later; return to theatre is needed to remove the blood, and haematoma can increase the risk of later capsular contracture.

source: guide-booklet

Record 20

topic: Infection

statement: Infection occurs in less than 1 in 1000 cases; most established infections around an implant require removal, with three to six months before a new implant can be inserted.

source: guide-booklet

Record 21

topic: Asymmetry

statement: Perfect symmetry is not an achievable goal; asymmetry can involve size, shape, nipple position and implant settling, and may appear more pronounced after surgery.

source: guide-booklet

Record 22

topic: Capsular contracture

statement: Noticeable firmness or capsular contracture occurs in up to 1 in 10 of all augmentations, but most of

those patients will not need revision surgery; re-operation for any reason is about 1% a year, around 10% after 10 years, with contracture the most common reason.

source: guide-booklet

Record 23

topic: Nipple sensation

statement: Around 1 in 5 patients will have reduced nipple sensation; recovery can take up to 12 months, but 1 in 10 patients will have some permanent numbness.

source: guide-booklet

Record 24

topic: Palpability

statement: It is common to be able to feel the implant, especially in slim patients or those with little breast tissue; this is an inevitable consequence of the operation and will not improve with time.

source: guide-booklet

Record 25

topic: Implant rupture

statement: The American Core study suggested 1 in 10 implants had ruptured at 10 years and half of all implants may rupture by 15 years; many ruptures are silent and contained within the capsule, but a symptomatic rupture should be investigated and the implant removed.

source: guide-booklet

Record 26

topic: Replacement schedule

statement: There is no universally agreed replacement schedule for breast implants and it is unusual to need exchange before ten years; routine replacement after a specified time is not recommended.

source: guide-booklet

Record 27

topic: BIA-ALCL

statement: BIA-ALCL is a rare cancer of the implant capsule recognised by the WHO, occurring 2 to 28 years after insertion (average 8 years), usually presenting as swelling around the implant, most often with textured implants — especially coarser textures — and usually treated by removing the implant and capsule.

source: guide-booklet

Record 28

topic: Breast Implant Illness

statement: Breast Implant Illness is a term some patients use for symptoms including tiredness, brain fog, joint aches and others; it is not a medical diagnosis and there is no proven association with implants, though some patients report improvement after removal.

source: guide-booklet

Record 29

topic: Mammography

statement: Implants interfere with mammography because X-rays cannot pass through them; tell the screening service you have implants so special views can be taken, and the more breast volume that is implant the greater the problem; some patients choose private MRI screening.

source: guide-booklet

Record 30

topic: Breastfeeding and pregnancy

statement: Implants do not interfere with the ability to breastfeed, though some evidence suggests milk production may be reduced; pregnancy and breastfeeding may adversely affect the shape of the breast.

source: guide-booklet

Record 31

topic: Myths

statement: It is safe to sunbathe and fly with implants; implant massage is not needed and is not recommended with modern textured implants.

source: guide-booklet

Record 32

topic: Fat transfer alternative

statement: Lipofilling is the only alternative surgical technique to enlarge the breast; only a relatively small amount of fat can be injected so multiple operations are needed, some fat is absorbed early, and fat that lasts brings permanent enlargement — your own fat is the only substance that can be safely injected.

source: guide-booklet

Record 33

topic: Uplift

statement: Mastopexy lifts the nipple and breast tissue and tightens skin without an implant; where both volume loss and droopiness are concerns, augmentation with mastopexy — as one operation or two stages — should be discussed.

source: guide-booklet

Record 34

topic: Implant registry

statement: The UK Breast and Cosmetic Implant Registry records all implants placed or removed so patients can be traced in a future recall; entry requires consent and is strongly recommended for anyone having implant-based surgery.

source: guide-booklet

Record 35

topic: Implant safety approval

statement: Breast implants continue to have safety approval from government organisations such as the UK MHRA and USA FDA and continue to be used in breast reconstruction worldwide.

source: implant-safety

Record 36

topic: BIA-SCC and other lymphomas

statement: In July 2023 BAAPS issued a joint statement on Breast Implant Associated Squamous Cell Carcinoma and other breast implant associated lymphomas.

source: implant-safety

Record 37

topic: PIP implants

statement: The 2013 expert group found no evidence PIP implants increased breast cancer rate, but the gel was less cohesive and had greater potential to cause local inflammation; BAAPS advised removal or replacement as a precaution.

source: implant-safety

Record 38

topic: PIP warning signs

statement: Signs of PIP rupture, leak or inflammation include breast lumpiness, lymph node swelling, change in shape or size, redness, tenderness, swelling and firmness; symptoms may include pain and hypersensitivity.

source: implant-safety

Record 39

topic: Consumer decision

statement: Only you can decide whether to go ahead with aesthetic surgery; no procedure is 100% risk free and no surgeon can give a 100% guarantee of results.

source: consumer-safety-guidelines

Record 40

topic: Cooling-off (consumer guidance)

statement: You should expect a two-week cooling-off period following your consultation, and should avoid booking fees, non-refundable advances and any technique that traps you into proceeding.

source: consumer-safety-guidelines

Record 41

topic: Surgeon credentials

statement: Your consultation should be with the surgeon who will operate, and a specialist plastic surgeon should have FCRS (Plast) and be on the GMC Specialist Register of Plastic Surgeons.

source: consumer-safety-guidelines

Record 42

topic: Timing and support

statement: Avoid surgery around major life events, consider travel and insurance implications carefully, talk to your GP, and take as much time as you need — you can cancel up until the time you go to sleep for surgery.

source: consumer-safety-guidelines

Research dataset: clevelandclinic-breast-enlargement

Cleveland Clinic — https://my.clevelandclinic.org/

generated: 2026-10-20

source: Cleveland Clinic

sourceUrl: https://my.clevelandclinic.org/

about: Cleveland Clinic Health Library patient information related to breast enlargement: breast augmentation, breast implants (types, placement, longevity), fat transfer breast augmentation, preparation, recovery, risks and complications.

categories

Record 1

id: augmentation

name: Breast augmentation

description: The Cleveland Clinic overview of breast augmentation: what it is, the two main types (implants and fat transfer), why people choose it, consultation, preparation, the operation itself and aftercare.

Record 2

id: implants

name: Breast implants

description: Cleveland Clinic information on breast implants themselves: who gets them, implant types, natural look and feel, sizing and placement, how long they last, advantages, risks and recovery.

Record 3

id: fat-transfer

name: Fat transfer breast augmentation

description: Cleveland Clinic information on using your own fat (liposuctioned and reinjected) to enlarge the breasts: candidacy, the procedure, how long results last, benefits, downsides, recovery and mammograms afterwards.

documents

Record 1

id: breast-augmentation

title: Breast Augmentation: What it is, Types, Surgery & Recovery

category: augmentation

kind: Treatment overview

url: https://my.clevelandclinic.org/health/treatments/11024-breast-augmentation

updated: 2021-08-24

summary: Cleveland Clinic's overview of breast augmentation, a common cosmetic surgery that uses breast implants or fat transfer to increase the size and shape of the breasts. It covers the implant types (saline, structured saline, silicone, form-stable, round, smooth, textured), fat transfer augmentation, why people choose

the procedure, how common it is, the consultation and preparation, the steps of surgery (anaesthesia, incision, implant placement, closing), what happens afterwards, benefits, side effects and complications, cancer screening with implants, recovery time, when to call the doctor, insurance, reversibility and breastfeeding.

keyPoints

- Breast augmentation is a surgical procedure that increases the size and shape of your breasts using breast implants or fat transfer.
- Two main types: breast implants and fat transfer augmentation; certain implants are only FDA-approved for certain ages.
- Saline implants are filled with sterile salt water; if they break, your body absorbs the saline naturally.
- Silicone implants are filled with silicone gel; if they break the gel can stay in the shell or leak into the breast, and regular checks with your surgeon may be needed.
- Form-stable ("gummy bear") implants keep their shape even if the shell breaks, are made of thicker gel and need a longer incision.
- Round implants make breasts look fuller and don't change the look of the breast if they rotate; smooth implants feel softest and move most naturally.
- Textured implants encourage scar tissue to adhere so they move less, but breast implant-associated anaplastic large cell lymphoma (BIA-ALCL) occurs most frequently with textured surfaces.
- In fat transfer augmentation, liposuction takes fat from the belly, flanks, back or thighs and injects it into the breasts; it is usually for a relatively small increase in size.
- People choose augmentation to change size, correct asymmetry, restore size after pregnancy or weight loss, correct unevenness after other breast surgery, or to improve confidence.
- Around 300,000 people in the United States have breast augmentation surgery every year; it is the most popular type of cosmetic surgery.
- At consultation your surgeon reviews medications, allergies, smoking, prior surgeries and breast history; you should have realistic expectations and be in good mental and physical health.
- Preparation may include a blood test, adjusting medications, stopping smoking, avoiding aspirin and anti-inflammatory drugs, and stopping recreational drugs.
- Arrange a driver and someone to stay the first night; take at least three days off work, or at least three weeks for a labour-intensive job.
- Surgery is done under general anaesthesia or IV sedation; incisions can be at the inframammary fold, around the areola (periareolar) or through the armpit (transaxillary).
- Implants are placed either under the breast tissue in front of the muscle or behind the pectoral muscle, depending on implant type and how much size is increased.
- Stitches are usually removed in about one week; no heavy lifting for at least four weeks and no sports for up to six weeks.
- Full recovery takes about six to eight weeks on average; recovery varies by augmentation type and overall health.
- Side effects include soreness, nipple sensitivity, bruising, bleeding and swelling; immediate complications include infection, wound healing issues and hematoma.
- Longer-term complications include capsular contracture (scar tissue), implant sagging, symmastia, seroma (fluid around the implant) and implant rupture or deflation.
- BIA-ALCL is a rare immune-system cancer reported with textured implants; symptoms such as size or shape changes, swelling or lumps may appear two to eight years after surgery.
- Breast implants are not guaranteed to last a lifetime; pregnancy, weight loss and menopause may change the appearance of augmented breasts.
- Implants can make mammograms harder to read; the American College of Radiology and related bodies recommend breast examinations at an ACR-accredited facility, which can take special views.
- Breast augmentation is usually considered elective cosmetic surgery, so insurance generally does not cover it;

reconstruction after cancer may be covered.

- Yes, augmentation is reversible: implants can be removed, replaced or repositioned by a plastic surgeon.
- Augmentation can affect nerves and ducts involved in lactation; implants placed below the muscle affect milk production less than those above, and periareolar incisions are more likely to reduce milk production.

Record 2

id: breast-implants

title: Breast Implants: Types, Surgery, Recovery & Risks

category: implants

kind: Treatment overview

url: https://my.clevelandclinic.org/health/treatments/21724-breast-implants

updated: 2021-08-31

summary: Cleveland Clinic's guide to breast implants: artificial silicone shells filled with silicone gel or saline, inserted by plastic surgeons for augmentation or reconstruction. It covers candidacy, reasons for getting implants, how common they are, how long implants last, which implants feel and look most natural, age limits for saline and silicone implants, sizing and placement, the four incision types, the procedure, recovery timeline, follow-up schedule, advantages, risks and the questions to ask your provider.

keyPoints

- Breast implants are artificial devices (prostheses): silicone shells filled with silicone gel or saline, inserted by plastic surgeons.
- Candidates should be finished developing, in good physical health and have realistic expectations.
- People get implants to restore previous breast size after pregnancy, weight loss or aging, to correct asymmetry, to restore breasts after mastectomy, or to boost confidence.
- About 3.5 million people in the United States have breast implants; roughly 60% contain silicone gel; about 35 million people worldwide have implants.
- Breast augmentation is the procedure that makes breasts larger; breast implants are the devices used in many augmentations.
- Implants typically last about 10 years; the older they get, the more likely they are to rupture; signs include changes in size, shape, firmness or swelling, though rupture can also have no signs.
- Silicone implants tend to feel more natural than saline; smooth shells feel more natural and move more freely than thicker textured shells.
- The FDA approved highly cohesive silicone gel ("gummy bear") implants in 2012; teardrop-shaped implants usually come with a textured shell so they don't rotate.
- You must be at least 18 years old for saline-filled implants and the FDA recommends at least 22 years old for silicone-filled implants.
- Providers assess mental health and social factors, including body dysmorphic disorder concerns and whether a partner is pressuring the decision.
- Implant size is matched to body structure: larger implants suit broader shoulders or wider hips, while oversized implants on a small frame can cause back, neck and shoulder pain.
- Preoperative sizing lets you try sample implants under clothing in front of a mirror before choosing size and shell type.
- Two placement options: submuscular (beneath chest muscles) and subglandular (between chest muscles and breast tissue).
- Four incision types: periareolar (around the areola), inframammary (breast crease), transaxillary (armpit) and transumbilical (belly button).
- After surgery breasts are wrapped in a supportive bra or compression band; you need a driver and someone to care for you for the first 24 hours.

- Full recovery is on average six to eight weeks; the first several days are the most painful, with swelling, bruising and slight incision bleeding.
- Pain usually decreases after five to seven days; light daily activities and desk work can resume after one to two weeks, but avoid running, heavy lifting and arms above head.
- Return to work is typically between two and six weeks, depending on how physically demanding the job is.
- Advantages: implants are durable (about 10 years), implant surgery has a relatively low risk of complications, and implants can boost self-esteem.
- Risks include short- and long-term complications, the need for additional surgery (removal, replacement, scar-tissue removal or repositioning), BIA-ALCL and breastfeeding complications.
- Implants increase size but do not lift; a breast lift can be combined if sagging is a concern.
- Follow-up: bandages and possibly stitches removed after one to two weeks, then reviews at four to six weeks, three months and six months.
- Ask which implant type and size suit your body, how long implants last, what happens on rupture, what autologous breast augmentation is, and the full list of risks.

Record 3

id: fat-transfer

title: Fat Transfer Breast Augmentation: Procedure, Results & Risks

category: fat-transfer

kind: Treatment overview

url: https://my.clevelandclinic.org/health/treatments/24033-fat-transfer-breast-augmentation

updated: 2022-08-18

summary: Cleveland Clinic's guide to fat transfer breast augmentation: liposuction removes fat from areas such as the belly, hips, back, legs or arms, and the purified fat is injected into the breasts to increase cup size by one to two sizes. It explains how the procedure differs from implants and reconstruction, who is and is not a candidate, how the outpatient surgery is performed, that results are lifelong (settling over up to six months), the risks, benefits and cons, recovery, when to call the doctor, and the effect on breast cancer risk and mammograms.

keyPoints

- A fat transfer breast augmentation uses fat from your own body to enlarge your breasts: fat is suctioned from several areas and injected into the breasts.
- Breast augmentation is cosmetic surgery to increase breast size and enhance shape; the two types are breast implants and fat transfer.
- Breast augmentation is for healthy people improving appearance and is not medically necessary; breast reconstruction after cancer always uses implants.
- Good candidates want slightly larger breasts without implants; people with little body fat, obesity, drastic weight fluctuations or who smoke may not be suitable.
- It is an outpatient procedure under general anaesthesia: fat is harvested by liposuction, purified in a centrifuge, then injected into different tissue layers of the breasts.
- Results are described as lifelong: fat cells settle over up to six months and remain for life, though extra fat is injected because some cells die after transfer.
- Risks include bleeding and bruising, fat cell death (necrosis) or fat migration, infection, and benign breast cysts or calcifications.
- Benefits over implants: a natural look and feel, the donor areas (belly, hips, back, legs or arms) become slimmer, and no implant-related risks such as rupture or capsular contracture.
- Cons: only one or two cup sizes of increase, breasts may still sag (a breast lift would be needed), and multiple procedures may be required.

- Recovery: chest wrapped with an elastic bandage and surgical bra, about a week off work, pain bruising and swelling for up to three weeks, swelling may last up to six months.
- Sleep on your back, take antibiotics and pain relievers as directed, and wear loose clothing to ease recovery.
- Call your provider for excessive bleeding, swelling or bruising, nausea and vomiting, severe pain, or signs of infection such as yellow discharge or fever.
- Fat transfer does not increase breast cancer risk and mammograms still work well afterwards, though it can cause microcalcifications that may need further tests such as ultrasound, MRI or needle biopsy.

facts

Record 1

topic: What breast augmentation is

statement: Breast augmentation is a common surgical procedure that increases the size and shape of your breasts using breast implants or fat transfer.

source: breast-augmentation

Record 2

topic: Two main types

statement: There are two main types of breast augmentation: breast implants and fat transfer augmentation, each with several options based on how you want your breasts to look and feel.

source: breast-augmentation

Record 3

topic: Saline implants

statement: Saline breast implants are filled with sterile salt water; if an implant breaks inside your breast, your body absorbs the saline and naturally gets rid of it.

source: breast-augmentation

Record 4

topic: Structured saline implants

statement: Structured saline implants are also filled with sterile saline but have an inner structure that helps the implant feel more natural.

source: breast-augmentation

Record 5

topic: Silicone implants

statement: Silicone breast implants are made of silicone gel; if the implant breaks the gel can stay within the shell or leak into the breast, and regular checks with your plastic surgeon may be required.

source: breast-augmentation

Record 6

topic: Form-stable implants

statement: Form-stable (gummy bear) implants keep their shape even if the shell breaks; they use a thicker silicone gel, are firmer than traditional implants and need a longer skin incision.

source: breast-augmentation

Record 7

topic: Round vs smooth vs textured

statement: Round implants make breasts look fuller and don't change the look of the breast if they rotate; smooth implants feel softest and move most naturally; textured implants adhere to scar tissue so they move less.

source: breast-augmentation

Record 8

topic: Textured implants and BIA-ALCL

statement: Breast implant-associated anaplastic large cell lymphoma (BIA-ALCL), though rare, occurs most frequently in people with breast implants that have textured surfaces.

source: breast-augmentation

Record 9

topic: Fat transfer augmentation

statement: In fat transfer augmentation, liposuction takes fat from the belly, flanks, back or thighs and injects it into the breasts; it is usually for people wanting a relatively small increase in breast size.

source: breast-augmentation

Record 10

topic: Reasons for augmentation

statement: People choose breast augmentation to change breast appearance, correct asymmetry, restore size after pregnancy or weight loss, correct unevenness after other breast surgery, or improve confidence and self-esteem.

source: breast-augmentation

Record 11

topic: How common it is

statement: Breast augmentation is the most popular type of cosmetic surgery; around 300,000 people in the United States have the surgery every year.

source: breast-augmentation

Record 12

topic: Consultation questions

statement: At consultation you should ask whether the surgeon is board-certified (American Board of Plastic Surgery), how often they perform breast augmentations, whether you can see before-and-after pictures, whether implants or fat transfer suit you, and what happens if you are not satisfied with the results.

source: breast-augmentation

Record 13

topic: Preparation

statement: Preparation may include a blood test, adjusting medications, stopping smoking, avoiding aspirin and certain anti-inflammatory drugs (which increase bleeding), and stopping recreational drugs.

source: breast-augmentation

Record 14

topic: Time off work

statement: You need someone to drive you home and stay with you the first night; take at least three days off work, or at least three weeks if you have a labour-intensive job.

source: breast-augmentation

Record 15

topic: Anaesthesia and incisions

statement: Surgery is performed under general anaesthesia or IV sedation; incisions can be made at the inframammary fold (crease under the breast), around the areola (periareolar) or through the armpit (transaxillary).

source: breast-augmentation

Record 16

topic: Implant placement

statement: Implants are placed either under the breast tissue in front of the muscle or behind the pectoral muscle; placement depends on implant type and how much size you are increasing.

source: breast-augmentation

Record 17

topic: After surgery

statement: After surgery you are observed while waking up (usually about an hour), given pain medication and follow-up instructions, and sent home with a gauze bandage and possibly a surgical bra; drainage tubes are removed as instructed.

source: breast-augmentation

Record 18

topic: Recovery timeline

statement: Stitches are usually removed in about one week; no heavy lifting for at least four weeks; sports may take up to six weeks; full recovery averages six to eight weeks.

source: breast-augmentation

Record 19

topic: Side effects

statement: Side effects can include soreness and increased nipple sensitivity, bruising, bleeding and swelling.

source: breast-augmentation

Record 20

topic: Immediate complications

statement: Immediate complications, though rare, can include infection, wound healing issues and hematoma (blood collection in the surgical area).

source: breast-augmentation

Record 21

topic: Long-term complications

statement: Long-term complications can include capsular contracture (scar tissue formation), implant sagging, symmastia (implants drifting toward the middle of the chest), seroma (fluid around the implant) and implant rupture or deflation.

source: breast-augmentation

Record 22

topic: Implants are not lifelong

statement: Breast implants are not guaranteed to last your entire life; you may need another procedure to replace one or both implants, and pregnancy, weight loss and menopause may affect the result.

source: breast-augmentation

Record 23

topic: Cancer screening

statement: Implants can make self-examination and mammograms more difficult; the American College of Radiology, American Cancer Society, American Society of Breast Imaging and American Society of Plastic and Reconstructive Surgeons recommend breast examinations at an ACR-accredited facility capable of special views.

source: breast-augmentation

Record 24

topic: Insurance

statement: Breast augmentation is usually considered elective cosmetic surgery, so insurance generally does not cover the procedure or follow-up visits; reconstruction after breast cancer may be covered.

source: breast-augmentation

Record 25

topic: Reversibility

statement: Breast augmentation is reversible: a plastic surgeon can remove, replace or reposition implants through breast implant removal surgery.

source: breast-augmentation

Record 26

topic: Breastfeeding

statement: Augmentation can affect nerves and ducts involved in lactation; implants placed below the muscle affect milk production less than those above it, and periareolar incisions are more likely to reduce milk production.

source: breast-augmentation

Record 27

topic: When to call the doctor

statement: Contact your healthcare provider immediately if you develop fever, abnormal discharge such as pus from the incision, rupture of your stitches, or increasing enlargement or firmness of a breast with pain and pressure.

source: breast-augmentation

Record 28

topic: What implants are

statement: Breast implants are artificial silicone shells filled with silicone gel or saline, inserted by plastic surgeons to change the size or shape of the breasts.

source: breast-implants

Record 29

topic: Implant prevalence

statement: About 3.5 million people in the United States have breast implants (roughly 60% silicone gel), and around 35 million people worldwide.

source: breast-implants

Record 30

topic: Implant longevity

statement: New breast implants typically last about 10 years; the older they are, the more likely they are to rupture, sometimes without any signs such as changes in size, shape, firmness or swelling.

source: breast-implants

Record 31

topic: Age limits

statement: You have to be at least 18 years old for saline-filled breast implants, and the FDA recommends being at least 22 years old for silicone-filled implants.

source: breast-implants

Record 32

topic: Sizing

statement: Larger implants often suit broader shoulders or wider hips, while larger implants on a smaller frame can cause back, neck and shoulder pain; preoperative sizing lets you try samples under clothing before deciding.

source: breast-implants

Record 33

topic: Natural look and feel

statement: Silicone implants tend to feel more natural than saline; submuscular placement means you only feel natural breast tissue on top of the implant, and smooth shells feel and move more naturally than textured shells.

source: breast-implants

Record 34

topic: Incision types

statement: Implant incisions can be periareolar (around the areola), inframammary (breast crease), transaxillary (armpit) or transumbilical (belly button).

source: breast-implants

Record 35

topic: Risks of implants

statement: Risks of breast implants include short- and long-term complications, additional surgery for removal, replacement, scar-tissue removal or repositioning, BIA-ALCL, and breastfeeding complications.

source: breast-implants

Record 36

topic: Implants do not lift

statement: Breast implants increase size but do not lift the breasts; if sagging is a concern, discuss combining a breast lift with implants.

source: breast-implants

Record 37

topic: Follow-up schedule

statement: After implant surgery, bandages and possibly stitches are removed at one to two weeks, with further reviews at four to six weeks, three months and six months.

source: breast-implants

Record 38

topic: Fat transfer basics

statement: Fat transfer breast augmentation suctions fat from areas such as the belly, hips, back, legs or arms and injects the purified cells into the breasts, so donor areas become slimmer while the breasts get bigger.

source: fat-transfer

Record 39

topic: Fat transfer candidacy

statement: People with little body fat may not be good candidates because more fat must be removed than used; people with obesity, drastic weight fluctuations or who smoke may also be unsuitable.

source: fat-transfer

Record 40

topic: Fat transfer procedure

statement: The procedure is an outpatient surgery under general anaesthesia: fat is harvested by liposuction, purified in a centrifuge, then injected into different tissue layers of the breasts.

source: fat-transfer

Record 41

topic: Fat transfer results

statement: Results from a fat transfer breast augmentation are described as lifelong; final results can take up to six months to appear as fat cells settle, and extra fat is injected because some cells die after transfer.

source: fat-transfer

Record 42

topic: Fat transfer limits

statement: Fat transfer increases breast size by only one or two cup sizes, does not fix sagging (a breast lift is needed for that), and may need multiple procedures to achieve the desired result.

source: fat-transfer

Record 43

topic: Fat transfer risks

statement: Risks of fat transfer include bleeding and bruising, fat cell death (necrosis) or fat migrating to other parts of the body, infection, and benign breast cysts or calcifications.

source: fat-transfer

Record 44

topic: Fat transfer recovery

statement: After fat transfer the chest is wrapped with an elastic bandage and surgical bra; plan about a week off work, with pain, bruising and swelling for up to three weeks and swelling that may persist up to six months.

source: fat-transfer

Record 45

topic: Cancer risk and mammograms

statement: A fat transfer breast augmentation does not increase breast cancer risk and mammograms still work well afterwards, though it can cause microcalcifications that sometimes need ultrasound, MRI or needle biopsy to rule out cancer.

source: fat-transfer

Research dataset: cochrane-breast-enlargement

Cochrane Library (Cochrane Database of Systematic Reviews) — https://www.cochranelibrary.com/

generated: 20 October 2026

source: Cochrane Library (Cochrane Database of Systematic Reviews)

sourceUrl: https://www.cochranelibrary.com/

searchUrl: https://www.cochranelibrary.com/search?searchBy=0&searchText=breast+augmentation

total: 7

reviews: 3

protocols: 4

categories

Record 1

id: augmentation

name: Augmentation and implant technique

description: Cochrane records that address breast enlargement directly — wound drains after augmentation surgery and the implant insertion techniques used to prevent capsular contracture.

Record 2

id: implant-types

name: Implant types and materials

description: Cochrane reviews comparing implant surfaces, fillers and shapes, and what the randomised evidence does and does not establish about choosing between them.

Record 3

id: reconstruction

name: Implant-based reconstruction

description: Reviews and registered protocols on reconstruction after mastectomy: implants versus the patient's own tissue, and the meshes and dermal matrices used to support implants.

Record 4

id: fat-grafting

name: Fat transfer

description: The Cochrane record on fat grafting — withdrawn before any review was completed — and the evidence gap it leaves.

documents

Record 1

id: CD007258.pub3

title: Wound drainage after plastic and reconstructive surgery of the breast

category: augmentation

kind: Cochrane Review

status: Completed review

updated: 21 October 2015

doi: 10.1002/14651858.CD007258.pub3

pmid: 26487173

url: https://www.cochranelibrary.com/cdsr/doi/10.1002/14651858.CD007258.pub3/full

doiUrl: https://doi.org/10.1002/14651858.CD007258.pub3

summary: Whether surgical wound drains should be placed after breast augmentation, reduction or reconstruction. Three randomised trials (306 women, 505 breasts) were found out of 190 screened studies — all of them in breast reduction — comparing drains with no drains after surgery.

keyPoints

- Across the three trials, drains produced no statistically significant improvement in infection, other wound complications, pain, bruising, seroma, haematoma or reoperation.
- Women who were not drained left hospital a mean 0.77 days sooner (95% CI 0.40 to 1.14 days sooner; low-certainty evidence) — the only significant result.
- No trials of drains after breast augmentation or breast reconstruction were identified, so the findings apply to breast reduction only.
- Cochrane concludes that the limited evidence shows no significant benefit of routine post-operative drains, and that augmentation and reconstruction still require investigation.

Record 2

id: CD010895.pub2

title: Different types of implants for reconstructive breast surgery

category: implant-types

kind: Cochrane Review

status: Completed review

updated: 16 May 2016

doi: 10.1002/14651858.CD010895.pub2

pmid: 27182693

url: https://www.cochranelibrary.com/cdsr/doi/10.1002/14651858.CD010895.pub2/full

doiUrl: https://doi.org/10.1002/14651858.CD010895.pub2

summary: Whether different breast implant types — textured or smooth, silicone or saline, anatomical or round, variable or fixed volume — change capsular contracture, complications, satisfaction and quality of life after reconstructive breast surgery. Five randomised trials with 202 participants were found.

keyPoints

- Every comparison was rated very low quality evidence: the trials were small, at unclear or high risk of bias, and too different from each other to pool.
- In one trial of 60 women, saline implants had less severe capsular contracture and higher satisfaction than silicone implants, but reinterventions were significantly more frequent with saline.
- No comparison reached statistical significance for textured versus smooth silicone implants (1 study, 20 women) or anatomical versus round implants (1 study, 36 women).
- No trials compared fifth-generation silicone implants or different manufacturers; the review concludes that

surgeons still cannot counsel women on implant choice on the basis of randomised evidence.

Record 3

id: CD013821.pub2

title: Implants versus autologous tissue flaps for breast reconstruction following mastectomy

category: reconstruction

kind: Cochrane Review

status: Completed review

updated: 31 October 2024

doi: 10.1002/14651858.CD013821.pub2

pmid: 39479986

url: https://www.cochranelibrary.com/cdsr/doi/10.1002/14651858.CD013821.pub2/full

doiUrl: https://doi.org/10.1002/14651858.CD013821.pub2

summary: Implant-based versus own-tissue (flap) reconstruction after mastectomy — quality of life, satisfaction and short- and long-term complications. Thirty-five non-randomised studies with 57,555 participants met the criteria; no randomised trials were found.

keyPoints

- Implants may reduce psychosocial well-being (mean difference -4.26 on a 0–100 scale; low-certainty evidence) and sexual well-being (-6.63; low-certainty) versus autologous flaps on BREAST-Q measures.
- Women may be less satisfied with the reconstruction with implants (-5.96; low-certainty) and with the breast itself (-8.17; very low-certainty).
- Long-term complications may be more frequent with implants (risk ratio 1.56, 95% CI 1.09 to 2.22; very low-certainty), while implant surgery was about 125 minutes shorter (very low-certainty).
- All evidence came from non-randomised studies (26 at serious overall risk of bias); the review found no superior technique for every woman and called for randomised trials.

Record 4

id: CD016344

title: Breast implant insertion funnels for preventing capsular contracture

category: augmentation

kind: Cochrane Protocol

status: Review in progress

updated: 30 March 2026

doi: 10.1002/14651858.CD016344

pmid: 41909996

url: https://www.cochranelibrary.com/cdsr/doi/10.1002/14651858.CD016344/full

doiUrl: https://doi.org/10.1002/14651858.CD016344

summary: Registered protocol for a review that will assess the effects of breast implant insertion funnels — devices used to guide the implant into place while minimising contact with skin — on capsular contracture rates in women having breast reconstruction or breast augmentation.

keyPoints

- This is a protocol, not a completed review: no results are available yet.
- The primary outcome planned is capsular contracture, alongside other complications, with both randomised and

non-randomised studies eligible.

- The protocol anticipates few, if any, randomised trials of funnels, because capsular contracture often appears years after surgery and has many causes.

Record 5

id: CD015961

title: Mesh for immediate implant-based prepectoral breast reconstruction following mastectomy

category: reconstruction

kind: Cochrane Protocol

status: Review in progress

updated: 17 June 2025

doi: 10.1002/14651858.CD015961

pmid: 40525534

url: https://www.cochranelibrary.com/cdsr/doi/10.1002/14651858.CD015961/full

doiUrl: https://doi.org/10.1002/14651858.CD015961

summary: Registered protocol for a review of biological or synthetic mesh used to support a prepectoral implant (placed above the muscle) in immediate breast reconstruction after mastectomy.

keyPoints

- This is a protocol, not a completed review: no results have been published.
- It plans to compare mesh versus no mesh, and biological versus synthetic mesh, for complications, patient-reported outcomes and reoperation after prepectoral reconstruction.
- Prepectoral placement is one of the placement options also discussed in breast augmentation, so the review will be relevant to implant positioning debate generally.

Record 6

id: CD011966

title: Acellular dermal matrices for breast reconstruction surgery

category: reconstruction

kind: Cochrane Protocol

status: Review in progress

updated: 2015

doi: 10.1002/14651858.CD011966

pmid:

url: https://www.cochranelibrary.com/cdsr/doi/10.1002/14651858.CD011966/full

doiUrl: https://doi.org/10.1002/14651858.CD011966

summary: Registered protocol for a review of acellular dermal matrices (biological meshes derived from human or animal skin) used around implants in immediate prosthetic breast reconstruction after cancer or risk-reducing surgery.

keyPoints

- This protocol was registered in 2015; no completed review has been published under this title.
- It plans to compare reconstruction with versus without acellular dermal matrix for seroma, infection, skin flap necrosis, haematoma, reconstruction failure and implant removal.

- The protocol notes that some studies suggest dermal matrix may raise complication rates, which the review is intended to test.

Record 7

id: CD010793.pub2

title: Autologous fat grafting for breast reconstruction

category: fat-grafting

kind: Cochrane Protocol

status: Withdrawn

updated: 29 June 2021

doi: 10.1002/14651858.CD010793.pub2

pmid:

url: https://www.cochranelibrary.com/cdsr/doi/10.1002/14651858.CD010793.pub2/full

doiUrl: https://doi.org/10.1002/14651858.CD010793.pub2

summary: Protocol for a review of autologous fat grafting — transferring the patient's own fat into the breast — for breast reconstruction after cancer surgery. Cochrane withdrew the protocol because the authors were no longer able to complete the review.

keyPoints

- Cochrane withdrew this protocol: "the authors are no longer able to complete this review", so no Cochrane systematic review of fat grafting exists under this title.
- The protocol itself was published on 29 June 2021 and remains readable as a record of the intended methods.
- No Cochrane review has examined fat transfer for cosmetic breast enlargement either.

facts

Record 1

topic: Wound drains

statement: Randomised evidence does not support routine drains after breast surgery: in three trials (306 women) drains did not reduce complications, and hospital stay was about 0.8 days shorter without them. Every trial was in breast reduction — none in augmentation.

source: CD007258.pub3

Record 2

topic: Implant type

statement: No randomised trial of usable quality establishes one implant type as better than another. All five trials (202 participants) were rated very low quality evidence, and neither textured-versus-smooth nor anatomical-versus-round comparisons reached significance.

source: CD010895.pub2

Record 3

topic: Implants versus own tissue

statement: Across 35 non-randomised studies (57,555 participants), implants may mean lower psychosocial and sexual well-being and less satisfaction than autologous flaps, but about 125 minutes shorter surgery — with low to very low certainty throughout.

source: CD013821.pub2

Record 4

topic: Capsular contracture

statement: Whether implant insertion funnels reduce capsular contracture is being assessed in a review registered in 2026. No results exist yet, and the protocol expects few randomised trials to be found.

source: CD016344

Record 5

topic: Mesh and dermal matrix

statement: Two registered protocols — mesh for prepectoral support and acellular dermal matrices — still have no completed review, leaving implant-support techniques without a Cochrane synthesis.

source: CD015961

Record 6

topic: Fat transfer

statement: Cochrane has never published a review of fat transfer for breast enlargement. The protocol on fat grafting for reconstruction was withdrawn in 2021 because the authors could not complete it.

source: CD010793.pub2

Research dataset: fda-breast-enlargement

U.S. Food and Drug Administration (FDA) — https://www.fda.gov/

generated: 2026-10-19

source: U.S. Food and Drug Administration (FDA)

sourceUrl: https://www.fda.gov/

about: FDA regulatory, safety and consumer information related to breast enlargement, breast augmentation, breast implants and injectable fillers promoted for breast enlargement.

categories

Record 1

id: implants-regulation

name: Breast implants — regulation and approval

description: How the FDA regulates saline and silicone gel-filled breast implants, which uses are approved, labeling requirements and restrictions on sale.

Record 2

id: implants-safety

name: Breast implants — safety and complications

description: Known risks, complications, rupture screening, revision surgery, monitoring and long-term follow-up for breast implants.

Record 3

id: cancers

name: BIA-ALCL and other cancers

description: Breast implant-associated anaplastic large cell lymphoma (BIA-ALCL), squamous cell carcinoma and other lymphomas reported in the capsule around implants.

Record 4

id: fillers

name: Dermal fillers and injectable silicone

description: Approved and unapproved uses of dermal fillers, the FDA position on fillers for breast enlargement, and warnings about injectable silicone.

Record 5

id: consumer

name: Consumer guidance and reporting

description: Patient-facing advice, decision checklists and how to report adverse events or suspected illegal products.

documents

Record 1

id: breast-implants-hub

title: Breast Implants

category: implants-regulation

kind: Regulatory hub

url: https://www.fda.gov/medical-devices/implants-and-prosthetics/breast-implants

updated: 2023-12-15

summary: The FDA's central breast implant resource. It explains that implants are medical devices placed under breast tissue or chest muscle to increase breast size (augmentation) or to replace tissue removed by cancer or trauma, and links to approvals, labeling, risks, safety communications and post-approval studies.

keyPoints

- Two implant types are approved for sale in the United States: saline-filled and silicone gel-filled. Both have a silicone outer shell.
- Implants vary in size, shell thickness, surface texture and shape.
- The FDA provides this information to support, not replace, the discussion between a patient and a physician.
- Patients are encouraged to report adverse events through MedWatch.

Record 2

id: types-of-implants

title: Types of Breast Implants

category: implants-regulation

kind: Regulatory page

url: https://www.fda.gov/medical-devices/breast-implants/types-breast-implants

updated: 2019-10-23

summary: Describes saline-filled and silicone gel-filled implants, their approved age groups, and the role of breast tissue expanders in reconstruction.

keyPoints

- Saline-filled implants are approved for breast augmentation in women aged 18 or older and for reconstruction at any age.
- Silicone gel-filled implants are approved for breast augmentation in women aged 22 or older and for reconstruction at any age.
- Breast implants are not lifetime devices; the longer they are in place, the more likely removal or replacement becomes.
- The most frequent complications are capsular contracture, reoperation and implant removal, with or without replacement.
- MRI remains the most effective method for detecting silent rupture; ultrasound is an acceptable alternative for screening asymptomatic patients.

Record 3

id: risks-complications

title: Risks and Complications of Breast Implants

category: implants-safety

kind: Regulatory page

url: https://www.fda.gov/medical-devices/breast-implants/risks-and-complications-breast-implants

updated: 2023-12-15

summary: A detailed list of local complications and adverse outcomes reported in at least one percent of breast implant patients, plus sections on reoperation, removal, capsular contracture, rupture, BIA-ALCL, systemic symptoms, breastfeeding and effects on children.

keyPoints

- Listed complications include asymmetry, breast pain, capsular contracture, deflation, delayed wound healing, extrusion, hematoma, infection, malposition, necrosis, sensation changes, rupture, seroma, wrinkling and unsatisfactory size or style.
- Capsular contracture is graded I–IV using the Baker scale; grades III and IV are considered severe and may need reoperation.
- Silicone gel ruptures are often silent; MRI is the most effective detection method and ultrasound is an acceptable alternative for asymptomatic screening.
- The FDA has not detected an association between silicone gel-filled implants and connective tissue disease, breast cancer or reproductive problems, but notes that larger and longer studies would be needed to rule these out.
- Some patients report systemic symptoms grouped under the term breast implant illness (BII); causes are poorly understood.
- Some women can breastfeed after augmentation and some cannot; implant removal without replacement can leave dimpling, concavity, puckering or sagging.

Record 4

id: things-to-consider

title: Things to Consider Before Getting Breast Implants

category: implants-safety

kind: Patient decision guide

url: https://www.fda.gov/medical-devices/breast-implants/things-consider-getting-breast-implants

updated: 2023-03-08

summary: The FDA's pre-surgery checklist covering informed consent, reoperation, monitoring, BIA-ALCL, BII, surgical mesh and the information patients should discuss with their surgeon.

keyPoints

- Patients should assume additional surgery may be needed because implants are not lifetime devices and complications can occur.
- Implant removal or replacement may not be covered by insurance, even when a complication occurs.
- The most common local complications are capsular contracture, reoperation and implant removal; others include rupture or deflation, wrinkling, asymmetry, scarring, pain and infection.
- BIA-ALCL is a cancer of the immune system, not breast cancer; it is treatable but can be fatal, and treatment usually involves removing the implant and capsule.
- Surgical mesh used in breast implant surgery has not been approved by the FDA for that use.
- Silicone implants may require regular MRI or ultrasound screening for silent rupture, which may not be covered by insurance.

Record 5

id: breast-implant-surgery

title: Breast Implant Surgery

category: implants-safety

kind: Regulatory page

url: https://www.fda.gov/medical-devices/breast-implants/breast-implant-surgery

updated: 2023-08-03

summary: Explains where breast implant surgery can be performed, the FDA position on surgical mesh, and questions patients should ask their surgeon.

keyPoints

- Breast implant surgery can be performed in a hospital or a surgery centre.
- The FDA has not cleared or approved any surgical mesh for use with breast implants or in breast reconstruction.
- Patients are advised to discuss the benefits and risks of mesh with their health care provider.

Record 6

id: labeling-guidance

title: Breast Implants — Certain Labeling Recommendations to Improve Patient Communication

category: implants-regulation

kind: FDA guidance (2020)

url: https://www.fda.gov/regulatory-information/search-fda-guidance-documents/breast-implants-certain-labeling-recommendations-improve-patient-communication

updated: 2020-09-29

summary: Final guidance recommending content and format for breast implant labeling, including a boxed warning, patient decision checklist, device description and rupture screening recommendations.

keyPoints

- Labeling should include a boxed warning and a patient decision checklist.
- Materials and device descriptions should list the types and quantities of chemicals and heavy metals found in or released by the implant.
- The guidance underpins the new labeling approved on 27 October 2021.
- The recommendations supplement the FDA's Saline, Silicone Gel, and Alternative Breast Implants guidance.

Record 7

id: saline-silicone-guidance

title: Saline, Silicone Gel, and Alternative Breast Implants

category: implants-regulation

kind: FDA guidance (2020)

url: https://www.fda.gov/regulatory-information/search-fda-guidance-documents/saline-silicone-gel-and-alternative-breast-implants

updated: 2020-09-29

summary: Guidance covering the information manufacturers should provide for saline, silicone gel and alternative breast implants, superseding the 2006 version.

keyPoints

- Applies to saline, silicone gel and alternative breast implants.
- Supersedes the previous version dated 17 November 2006.
- Should be read alongside the labeling recommendations guidance.

Record 8

id: safety-requirements-2021

title: FDA Strengthens Safety Requirements and Updates Study Results for Breast Implants

category: implants-regulation

kind: Press announcement

url: https://www.fda.gov/news-events/press-announcements/fda-strengthens-safety-requirements-and-updates-study-results-breast-implants

updated: 2021-10-27

summary: Announcement of orders restricting the sale and distribution of breast implants and approval of new labeling with a boxed warning, patient decision checklist, rupture screening recommendations and a patient device card.

keyPoints

- Sale and distribution are restricted to providers and facilities that give patients the approved risk information.
- The patient decision checklist must be reviewed, initialled and signed by the patient and signed by the implanting physician.
- New labeling includes a device description listing specific materials in the device.
- The FDA also released updated information on manufacturer post-approval studies.

Record 9

id: postmarket-safety

title: Breast Implant Postmarket Safety Information

category: implants-safety

kind: Regulatory page

url: https://www.fda.gov/medical-devices/breast-implants/breast-implant-postmarket-safety-information

updated: 2025-06-02

summary: Describes the FDA's ongoing postmarket review of breast implant safety, including medical device reports, BIA-ALCL, systemic symptoms and manufacturer post-approval studies.

keyPoints

- The FDA continually evaluates postmarket safety and manufacturer compliance with approval conditions.
- Review includes medical device reports for BIA-ALCL and systemic symptoms in women with breast implants.
- Ongoing post-approval studies are listed by manufacturer and application number.
- In July 2019 Allergan voluntarily recalled Natrelle BIOCELL textured breast implants and tissue expanders.

Record 10

id: bia-alcl-qa

title: Questions and Answers about Breast Implant-Associated Anaplastic Large Cell Lymphoma (BIA-ALCL)

category: cancers

kind: Patient and clinician Q&A

url: https://www.fda.gov/medical-devices/breast-implants/questions-and-answers-about-breast-implant-associated-anaplastic-large-cell-lymphoma-bia-alcl

updated: 2019-10-23

summary: FDA answers on BIA-ALCL: what it is, who is at risk, symptoms, diagnosis, treatment and what patients and clinicians should do.

keyPoints

- BIA-ALCL is a T-cell lymphoma of the immune system, not breast cancer.
- It is usually found in the fluid or scar tissue near the implant and may spread in some cases.
- Risk is higher with textured-surface implants than with smooth-surface implants.
- Main symptoms are persistent swelling, a mass or pain around the implant, often years after surgery.
- Most patients are treated successfully with surgery to remove the implant and surrounding capsule; some also need chemotherapy or radiation.
- The FDA does not recommend routine removal of implants in patients without symptoms.

Record 11

id: scc-safety-communication

title: UPDATE: Reports of Squamous Cell Carcinoma (SCC) in the Capsule Around Breast Implants

category: cancers

kind: Safety communication

url: https://www.fda.gov/medical-devices/safety-communications/update-reports-squamous-cell-carcinoma-scc-capsule-around-breast-implants-fda-safety-communication

updated: 2023-03-08

summary: Updated safety communication on reports of squamous cell carcinoma and various lymphomas in the scar capsule around breast implants.

keyPoints

- The communication updates a September 2022 safety communication.
- The lymphomas described are not the same as BIA-ALCL.
- The FDA believes cases of SCC and lymphomas in the capsule may be rare.
- Health care providers should discuss these risks with women who have or are considering breast implants.
- Patients and providers are asked to report cancers around breast implants to the FDA.

Record 12

id: dermal-fillers

title: Dermal Fillers (Soft Tissue Fillers)

category: fillers

kind: Regulatory page

url: https://www.fda.gov/medical-devices/aesthetic-cosmetic-devices/dermal-fillers-soft-tissue-fillers

updated: 2023-07-06

summary: Describes approved facial and hand uses of dermal fillers, the risks, and the FDA's position on unapproved body uses including breast enlargement.

keyPoints

- Dermal fillers are approved for specific facial uses, lip and cheek augmentation, back-of-hand augmentation, HIV lipoatrophy and contour deficiencies.
- The FDA recommends against using dermal fillers or any injectable filler to increase breast size (breast augmentation) or for other body contouring.
- These uses are not approved by the FDA, and the risks of unapproved uses or unapproved products are not known.
- Injectable silicone is not approved for any aesthetic procedure and can cause long-term pain, infection, scarring, embolism, stroke and death.
- Needle-free devices are not approved for injecting dermal fillers, and no dermal filler is approved for over-the-

counter use.

- Removal of filler may require injections, surgery or other interventions, and it may be difficult or impossible to remove permanent fillers.

Record 13

id: approved-fillers

title: FDA-Approved Dermal Fillers

category: fillers

kind: Regulatory list

url: https://www.fda.gov/medical-devices/aesthetic-cosmetic-devices/fda-approved-dermal-fillers

updated:

summary: The FDA's list of approved dermal fillers and their approved indications, limited to facial tissue and the hands.

keyPoints

- The list covers approved facial and hand products.
- The FDA has not approved liquid silicone or silicone gel for injection.
- Approved indications vary by product; not every product is reviewed for every body site.

Record 14

id: injectable-silicone-warning

title: FDA Warns About Illegal Use of Injectable Silicone for Body Contouring and Associated Health Risks

category: fillers

kind: Press announcement and safety communication

url: https://www.fda.gov/news-events/press-announcements/fda-warns-about-illegal-use-injectable-silicone-body-contouring-and-associated-health-risks

updated: 2017-11-13

summary: Warning about injectable silicone and products falsely marketed as FDA-approved fillers to enlarge the buttocks, breasts and other body parts.

keyPoints

- Injectable silicone used for body contouring is not FDA-approved.
- Serious side effects include ongoing pain, scarring, tissue death, permanent disfigurement, embolism, stroke, infection and death.
- Complications may occur immediately or weeks, months or years later.
- Silicone injections for body contouring are often performed by unlicensed providers in non-clinical settings.
- Injectable silicone differs from the silicone contained inside approved breast implants, because the implant shell keeps the gel from migrating.

Record 15

id: consumer-breast-implants

title: What to Know About Breast Implants

category: consumer

kind: Consumer update

url: https://www.fda.gov/consumers/consumer-updates/what-know-about-breast-implants

updated: 2025-01-31

summary: Patient-facing overview of implant types, risks, labeling, long-term monitoring, BIA-ALCL and communication with a surgeon.

keyPoints

- The FDA has approved implants for augmentation, reconstruction after cancer or trauma, correction of developmental defects, and revision of previous surgery.
- Risks include additional surgery, BIA-ALCL, systemic symptoms (BII), capsular contracture, breast pain, rupture, deflation, silent rupture and infection.
- Breast implants are not lifetime devices and their lifespan cannot be predicted.
- Mandatory labeling includes a patient decision checklist, boxed warning and patient device card.
- Patients with silicone implants are advised to have regular screening for silent rupture and to tell mammography facilities that they have implants.
- Injectable silicone is not approved for breast augmentation, reconstruction or body contouring.

Record 16

id: medwatch

title: MedWatch: FDA Safety Information and Adverse Event Reporting Program

category: consumer

kind: Reporting program

url: https://www.fda.gov/safety/medwatch-fda-safety-information-and-adverse-event-reporting-program

updated:

summary: How patients and clinicians report injuries, adverse events or product problems related to breast implants, fillers and other FDA-regulated products.

keyPoints

- Voluntary reports can be filed online or by phone at 1-800-FDA-1088.
- Reports help the FDA monitor approved products after they are sold.
- Useful details include device name, manufacturer and a description of the event and any interventions.

facts

Record 1

topic: Implants are devices, not procedures

statement: The FDA regulates breast implants as medical devices and reviews their safety and effectiveness; it does not approve breast enlargement procedures or clinics.

source: breast-implants-hub

Record 2

topic: Approved implant types

statement: Only saline-filled and silicone gel-filled breast implants are approved for sale in the United States. Both have a silicone outer shell.

source: breast-implants-hub

Record 3

topic: Age indications for augmentation

statement: Saline-filled implants are approved for breast augmentation from age 18; silicone gel-filled implants

are approved for augmentation from age 22. Both are approved for reconstruction at any age.

source: types-of-implants

Record 4

topic: Implants are not lifetime devices

statement: The longer implants remain in place, the more likely it is that they will need to be removed or replaced, and patients should assume further surgery may be needed.

source: things-to-consider

Record 5

topic: Most frequent complications

statement: The most common local complications are capsular contracture, reoperation and implant removal; others include rupture or deflation, wrinkling, asymmetry, scarring, pain and infection.

source: things-to-consider

Record 6

topic: Silent rupture

statement: Most silicone gel implant ruptures have no symptoms. MRI is the most effective detection method, and ultrasound is acceptable for screening asymptomatic patients.

source: risks-complications

Record 7

topic: BIA-ALCL

statement: BIA-ALCL is a T-cell lymphoma of the immune system, not breast cancer. It is more common with textured implants, often presents as persistent swelling, a mass or pain, and is usually treated by removing the implant and capsule.

source: bia-alcl-qa

Record 8

topic: SCC and other lymphomas

statement: The FDA has received reports of squamous cell carcinoma and various lymphomas in the scar capsule around breast implants. These are not the same as BIA-ALCL and are believed to be rare.

source: scc-safety-communication

Record 9

topic: Breast implant illness

statement: Some patients report systemic symptoms such as fatigue, memory loss, rash, brain fog and joint pain, collectively called breast implant illness. The causes are poorly understood and research is ongoing.

source: risks-complications

Record 10

topic: Fillers are not approved for breast enlargement

statement: The FDA recommends against using dermal fillers or any injectable filler to increase breast size. The use is not approved, and the risks of unapproved uses and products are not known.

source: dermal-fillers

Record 11

topic: Injectable silicone

statement: Injectable silicone is not approved for any aesthetic procedure. It can cause chronic pain, infection, scarring, permanent disfigurement, embolism, stroke and death.

source: dermal-fillers

Record 12

topic: Filler removal

statement: Removing or reducing filler may require injections, surgery or other interventions, and permanent or non-hyaluronic-acid fillers may be difficult or impossible to remove.

source: dermal-fillers

Record 13

topic: Labeling and consent

statement: Since 2021, FDA-approved breast implants carry a boxed warning, a patient decision checklist and a patient device card, and their sale is restricted to providers who give patients this information.

source: safety-requirements-2021

Record 14

topic: Surgical mesh

statement: The FDA has not cleared or approved any surgical mesh for use with breast implants or in breast reconstruction.

source: breast-implant-surgery

Record 15

topic: Reporting problems

statement: Patients and clinicians can report adverse events or product problems to the FDA through MedWatch online or by phone at 1-800-FDA-1088.

source: medwatch

Research dataset: healthdirect-breast-enlargement

healthdirect (Australian Government) — https://www.healthdirect.gov.au/

generated: 2026-10-20

source: healthdirect (Australian Government)

sourceUrl: https://www.healthdirect.gov.au/

about: healthdirect patient information on breast augmentation (breast implants): what the procedure involves, benefits and considerations, preparation and choosing a surgeon, costs and Medicare, recovery, complications, warning signs, alternatives and Australian support resources.

reviewed: November 2025

categories

Record 1

id: overview

name: What breast augmentation is

description: How healthdirect describes the procedure itself, why people choose it, and the points it says to weigh up before deciding.

Record 2

id: procedure

name: The procedure and preparation

description: Anaesthetic and implant placement, how to prepare for surgery, choosing a surgeon through AHPRA and RACS, what Medicare covers, and the questions healthdirect suggests asking before surgery.

Record 3

id: recovery

name: Recovery and warning signs

description: What to expect after surgery, the support typically needed while healing, and the symptoms that mean you should contact your surgeon or go to hospital.

Record 4

id: risks

name: Complications and risks

description: The possible problems healthdirect lists after breast implant surgery, from bleeding and infection to rarer serious risks.

Record 5

id: alternatives

name: Alternatives and Australian resources

description: Options other than implants, plus the Australian organisations, fact sheets and helplines healthdirect points readers to.

documents

Record 1

id: what-is

title: What is breast augmentation?

category: overview

kind: Procedure page section

url: https://www.healthdirect.gov.au/procedures/surgery/breast-augmentation-breast-implants#what-is

updated: November 2025

summary: healthdirect defines breast augmentation as a form of cosmetic surgery that enlarges the breasts, also called breast enlargement or breast reconstruction. Implants sit either under the breast or under the chest muscle and consist of a silicone shell filled with saline (salty water) or silicone gel. Some surgeons also offer fat grafting, moving fat from another part of the body into the breast. The page notes that reconstruction after mastectomy is a separate topic, and that a Therapeutic Goods Administration (TGA) review led to some breast implants being withdrawn from the Australian market or given conditions on their use.

keyPoints

- Breast augmentation is cosmetic surgery to make the breasts larger; it is also referred to as breast enlargement.
- Implants are placed under the breast or beneath the chest muscle and are made of a silicone shell filled with saline or silicone gel.
- Some cosmetic surgeons offer fat grafting, transferring fat from areas such as the buttocks into the breasts.
- Reconstruction after a mastectomy uses implants or tissue for a different purpose; that is separate from cosmetic augmentation.
- After a TGA review, some breast implant products have been removed from the Australian market or had conditions placed on them.

Record 2

id: benefits

title: What are the benefits of breast augmentation?

category: overview

kind: Procedure page section

url: https://www.healthdirect.gov.au/procedures/surgery/breast-augmentation-breast-implants#benefits

updated: November 2025

summary: healthdirect states that people consider breast augmentation for many reasons, most commonly a desire to feel more physically attractive or more comfortable with themselves. The page frames the benefit in terms of how the person hopes to feel rather than promising any particular result.

keyPoints

- Reasons people give include wanting to feel more physically attractive or better about themselves.
- healthdirect does not guarantee an outcome; expectations should be discussed with a surgeon.

Record 3

id: consider

title: What should I consider before choosing to have breast augmentation?

category: overview

kind: Procedure page section

url: https://www.healthdirect.gov.au/procedures/surgery/breast-augmentation-breast-implants#consider

updated: November 2025

summary: healthdirect acknowledges social pressure around body shape and asks readers to weigh several points before deciding: implants are not lifelong and usually need replacing after 10 to 15 years; cancer has very rarely arisen in the fluid and scar tissue around an implant; some people are unhappy with how their breasts look afterwards; implants can make breast cancer harder to detect during screening; and breastfeeding may be affected.

keyPoints

- Implants do not last for life and usually need replacing after 10 to 15 years.
- There have been rare cases of cancer developing in the fluid and scar tissue around an implant.
- Some people find the result does not look the way they hoped.
- Implants can make it harder to detect breast cancer during screening checks.
- The ability to breastfeed may change after surgery.
- Body image and social pressure are worth examining before deciding, and counselling is an option.

Record 4

id: involve

title: What does a breast augmentation procedure involve?

category: procedure

kind: Procedure page section

url: https://www.healthdirect.gov.au/procedures/surgery/breast-augmentation-breast-implants#involve

updated: November 2025

summary: The procedure is normally done under general anaesthetic so the patient sleeps throughout. The surgeon cuts alongside the breast and positions the implant either between the breast tissue and the muscle, or behind the large chest muscle under the breast; the surgeon decides which method suits the patient. Different surgical techniques are available, and the method chosen can affect recovery time.

keyPoints

- General anaesthetic is usually required, so you are asleep during the operation.
- The surgeon makes a cut alongside the breast.
- The implant is placed either between the breast tissue and the muscle, or behind the chest muscle.
- The surgeon chooses the method, and the technique used can affect how long recovery takes.

Record 5

id: prepare

title: How can I prepare for a breast augmentation?

category: procedure

kind: Procedure page section

url: https://www.healthdirect.gov.au/procedures/surgery/breast-augmentation-breast-implants#prepare

updated: November 2025

summary: healthdirect advises gathering as much information as possible and preparing by getting physically fit, reaching a healthy weight, trying to stop smoking, asking about medication changes, disclosing health conditions and allergies, and confirming whether you will stay overnight. On choosing a surgeon, it says to pick someone you feel comfortable with who will be honest about benefits and risks, notes that a GP must refer cosmetic surgery patients to their surgeon, and points to the AHPRA register where you can confirm the

surgeon is a Fellow of the Royal Australasian College of Surgeons (RACS) and check for serious disciplinary action. On cost, Medicare generally does not cover cosmetic surgery, private health insurance should be checked, and fees typically include the surgeon, anaesthetist, clinic or hospital, the implants, and medication, dressings and support garments. The page lists questions to ask about implant type, sensation, nipple position and appearance, bruising and pain, plus long-term questions about implant lifespan, breastfeeding, rupture, the likelihood of further operations and the cost of any repeat surgery.

keyPoints

- Prepare by getting physically fit, maintaining a healthy weight and trying to quit smoking.
- Ask whether any medications need to change, tell the surgeon about health conditions and allergies, and confirm whether you will stay overnight or go home.
- Choose a surgeon you feel comfortable with who will discuss benefits and risks honestly.
- A GP needs to make the referral to the surgeon you choose for cosmetic surgery.
- Check the AHPRA register to confirm your surgeon is a Fellow of the Royal Australasian College of Surgeons (RACS) and to see whether serious disciplinary action has been taken.
- Medicare generally does not cover cosmetic surgery; check your private health policy for any cover.
- Costs usually include surgeon and anaesthetist fees, clinic or hospital fees, the implants, and medication, dressings and support garments.
- Ask what implant type is recommended and why, whether sensation will change, how the nipples will be affected, and what bruising, swelling and pain to expect.
- Ask how long implants last, whether you will be able to breastfeed, how many of the surgeon's patients have had a ruptured or failed implant, how likely further operations are, and whether extra surgery would cost more.

Record 6

id: recover

title: How soon will I recover?

category: recovery

kind: Procedure page section

url: https://www.healthdirect.gov.au/procedures/surgery/breast-augmentation-breast-implants#recover

updated: November 2025

summary: Recovery depends on your health before surgery and your surgeon's advice; you may be asked to take a week off work and avoid moderate exercise for around five weeks. Tubes draining fluid, dressings and a compression bandage are common, along with pain and discomfort from swelling and bruising. Pain relief, extra support garments, time off work, help at home and a follow-up visit are often needed. healthdirect says to contact your surgeon or go to a hospital emergency department if you develop fever, severe pain, nausea or vomiting during recovery.

keyPoints

- Recovery varies with your pre-surgery health and your surgeon's advice.
- You may be asked to take about a week off work and avoid moderate exercise for around five weeks.
- Drain tubes, dressings and a compression bandage may be used, and pain, swelling and bruising are normal early on.
- You may need pain relief, support garments for a few weeks, time off work, help with daily tasks and a follow-up appointment.
- Fever, severe pain, nausea or vomiting during recovery means you should contact your surgeon or go to a hospital emergency department.

Record 7

id: complications

title: What complications can happen?

category: risks

kind: Procedure page section

url: https://www.healthdirect.gov.au/procedures/surgery/breast-augmentation-breast-implants#complications

updated: November 2025

summary: Like any operation, breast implant surgery carries risks that should be discussed in detail with a surgeon. healthdirect lists bleeding or a haematoma, infection, changed sensation or numbness in the breast skin or nipple, swelling or a fluid collection, injury to surrounding tissue and implant rupture. Rarer serious risks include an allergic reaction to the anaesthetic, blood clots in the legs that can travel to the lungs, and breast implant-associated cancer — implants previously linked with cancer are no longer available. Severe chest pain or trouble breathing means calling triple zero (000) for an ambulance.

keyPoints

- Possible problems include bleeding or a haematoma, infection, altered sensation or numbness, swelling or fluid collections, injury to nearby tissue and implant rupture.
- Rarer serious risks include allergic reaction to the anaesthetic and blood clots in the legs that can travel to the lungs.
- Breast implant-associated cancer is listed as a rare serious risk; implants linked with cancer in the past are no longer available.
- Risks should be discussed in detail with your surgeon before you consent.
- Trouble breathing or severe chest pain requires calling triple zero (000) for an ambulance.

Record 8

id: doctor

title: When should I see my doctor?

category: recovery

kind: Procedure page section

url: https://www.healthdirect.gov.au/procedures/surgery/breast-augmentation-breast-implants#doctor

updated: November 2025

summary: healthdirect lists the symptoms that mean contacting your surgeon straight away after surgery: a temperature above 38°C or chills; nausea, vomiting, shortness of breath or diarrhoea; heavy bleeding from the wound; bleeding or fluid leakage that starts more than a day after surgery; redness around the implant that is worsening or spreading; and increasing pain or tenderness. In an emergency, go to hospital or call triple zero (000).

keyPoints

- Contact your surgeon straight away for a temperature over 38°C or chills.
- Nausea, vomiting, shortness of breath or diarrhoea after surgery need urgent review.
- Heavy bleeding from the wound, or bleeding or fluid leakage starting more than one day after surgery, needs urgent review.
- Redness around the implant that worsens or spreads, and increasing pain or tenderness, should be reported.
- Fever, vomiting or heavy bleeding also warrants going to your nearest hospital.

Record 9

id: alternatives

title: Are there alternatives to breast augmentation?

category: alternatives

kind: Procedure page section

url: https://www.healthdirect.gov.au/procedures/surgery/breast-augmentation-breast-implants#alternatives

updated: November 2025

summary: healthdirect states that breast augmentation may not be right for everyone and lists counselling to help with accepting your body shape, wearing a padded bra, and other types of surgery that do not involve implants as alternatives.

keyPoints

- Counselling to help you accept your body shape is offered as an alternative.
- Wearing a padded bra is a non-surgical alternative.
- Other surgery that does not use implants may also be an option.

Record 10

id: resources

title: Resources and support

category: alternatives

kind: Procedure page section

url: https://www.healthdirect.gov.au/procedures/surgery/breast-augmentation-breast-implants#resources

updated: November 2025

summary: healthdirect recommends discussing the decision with your doctor first and links to the Australian Society of Plastic Surgeons, the Agency for Clinical Innovation breast implant safety patient information sheet, an Australian Government fact sheet on cosmetic surgery, and the TGA guide to the five questions to ask before getting a medical implant (available in community languages). It also gives the healthdirect helpline on 1800 022 222 (NURSE-ON-CALL in Victoria), staffed by registered nurses 24 hours a day, seven days a week.

keyPoints

- Discuss the decision with your doctor before going further.
- The Australian Society of Plastic Surgeons and the Agency for Clinical Innovation publish patient information on breast implants.
- The Australian Government has a cosmetic surgery fact sheet, and the TGA publishes a guide to questions to ask before a medical implant.
- The healthdirect helpline (1800 022 222, NURSE-ON-CALL in Victoria) is staffed by registered nurses 24/7.

Record 11

id: tga-status

title: Current status of breast implant products in Australia

category: overview

kind: Regulator callout on the healthdirect page

url: https://www.tga.gov.au/safety/safety-monitoring-and-information/safety-alerts/current-status-breast-implant-products-australia

updated: November 2025

summary: The healthdirect page carries a callout about a Therapeutic Goods Administration (TGA) review after which some breast implants were removed from the Australian market or had conditions placed on them, and links to the TGA's current status page for the details.

keyPoints

- The TGA reviewed breast implant products and some were withdrawn from the Australian market or given conditions of use.
- The TGA website publishes the current status of breast implant products in Australia.
- The TGA also publishes information on breast implants and anaplastic large cell lymphoma (BIA-ALCL).

facts

Record 1

topic: What the procedure is

statement: Breast augmentation is a form of cosmetic surgery that enlarges the breasts; it is also called breast enlargement, and uses implants placed under or behind the breast.

source: what-is

Record 2

topic: Implant construction

statement: Breast implants are a silicone shell filled with either saline (salty water) or silicone gel, positioned under the breast or beneath the chest muscle.

source: what-is

Record 3

topic: Fat grafting

statement: Some cosmetic surgeons also offer fat grafting, in which fat is taken from another part of the body, such as the buttocks, and transferred into the breasts.

source: what-is

Record 4

topic: TGA market action

statement: Following a Therapeutic Goods Administration review, some breast implants have been removed from the Australian market or had conditions placed on them; current status is published on the TGA website.

source: tga-status

Record 5

topic: Reasons people choose it

statement: People consider breast augmentation for many reasons, most often to feel more physically attractive or better about themselves.

source: benefits

Record 6

topic: Implant longevity

statement: Breast implants do not last for life and usually need to be replaced after 10 to 15 years.

source: consider

Record 7

topic: Rare cancer risk

statement: There have been rare cases of cancer growing in the fluid and scar tissue around a breast implant.

source: consider

Record 8

topic: Screening

statement: Breast implants can make breast cancer harder to detect during screening checks, so continuing screening remains important.

source: consider

Record 9

topic: Breastfeeding

statement: The ability to breastfeed may change after breast augmentation surgery.

source: consider

Record 10

topic: Satisfaction

statement: Some people find that after surgery their breasts do not look the way they hoped, which is a reason to discuss expectations carefully before consenting.

source: consider

Record 11

topic: Anaesthetic

statement: Breast augmentation normally requires a general anaesthetic, so the patient is asleep during the operation.

source: involve

Record 12

topic: Implant placement

statement: The implant is placed either between the breast tissue and the muscle or behind the large chest muscle under the breast, and the surgeon decides which method is right for you; the technique chosen can affect recovery time.

source: involve

Record 13

topic: Preparation

statement: Before surgery, healthdirect advises getting physically fit, having a healthy body weight, trying to quit smoking, asking about medication changes, disclosing health conditions and allergies, and confirming whether you will stay overnight.

source: prepare

Record 14

topic: Choosing a surgeon

statement: healthdirect advises choosing a surgeon you feel comfortable with who will speak honestly about benefits and risks, and verifying credentials on the AHPRA register of medical practitioners.

source: prepare

Record 15

topic: GP referral

statement: Your general practitioner needs to make the referral to the surgeon you choose for cosmetic surgery, and your GP can help you decide.

source: prepare

Record 16

topic: Surgeon credentials

statement: On the AHPRA register you can check that your surgeon is a Fellow of the Royal Australasian College of Surgeons (RACS) and see whether serious disciplinary action has been taken against them.

source: prepare

Record 17

topic: Medicare and insurance

statement: Medicare generally does not cover the cost of cosmetic surgery; if you have private health insurance you should check your policy to see whether it covers the procedure.

source: prepare

Record 18

topic: What the cost includes

statement: The cost of a breast augmentation varies by patient and typically includes surgeon and anaesthetist fees, clinic or hospital fees, the implants, and medication, dressings and support garments, all of which should be discussed with your surgeon beforehand.

source: prepare

Record 19

topic: Questions to ask

statement: healthdirect suggests asking which implant type is recommended and why, whether sensation will change, how the position and appearance of the nipples will be affected, and how much bruising, swelling and pain to expect.

source: prepare

Record 20

topic: Long-term questions

statement: Long-term questions to ask include how long the implants last, whether you will be able to breastfeed, how many patients the surgeon has seen with a ruptured or failed implant, how likely further operations are, and whether further surgery would cost extra.

source: prepare

Record 21

topic: Recovery time

statement: Recovery depends on your health before surgery and your surgeon's advice; you may be asked to take about a week off work and avoid moderate exercise for five weeks.

source: recover

Record 22

topic: After surgery

statement: After the operation you might have drain tubes, dressings, a compression bandage and pain and discomfort from swelling and bruising, and may need pain relief, support garments, help with daily activities and a follow-up appointment.

source: recover

Record 23

topic: Complications

statement: Possible complications include bleeding or a haematoma, infection, changed sensation or numbness in the breast skin or nipple, swelling or fluid collection, injury to surrounding tissue and implant rupture.

source: complications

Record 24

topic: Serious risks

statement: Rarer serious risks include allergic reaction to the anaesthetic, blood clots in the legs that can travel to the lungs, and breast implant-associated cancer; implants previously linked with cancer are no longer available.

source: complications

Record 25

topic: Emergency symptoms

statement: Trouble breathing or severe chest pain after surgery means calling triple zero (000) and asking for an ambulance.

source: complications

Record 26

topic: When to call your surgeon

statement: Contact your surgeon straight away for a temperature over 38°C or chills, nausea, vomiting, shortness of breath or diarrhoea, heavy bleeding, bleeding or fluid leakage starting more than a day after surgery, worsening or spreading redness around the implant, or increasing pain or tenderness.

source: doctor

Record 27

topic: Recovery emergencies

statement: During recovery, fever, severe pain, nausea or vomiting mean you should contact your surgeon or go to a hospital emergency department.

source: recover

Record 28

topic: Alternatives

statement: Alternatives to breast augmentation listed by healthdirect are counselling to help you accept your body shape, wearing a padded bra, and other types of surgery that do not involve implants.

source: alternatives

Record 29

topic: Australian resources

statement: healthdirect points to the Australian Society of Plastic Surgeons, the Agency for Clinical Innovation breast implant safety sheet, an Australian Government cosmetic surgery fact sheet and the TGA's guide to questions to ask before a medical implant.

source: resources

Record 30

topic: Helpline

statement: The healthdirect helpline on 1800 022 222 (NURSE-ON-CALL in Victoria) connects you with a registered nurse 24 hours a day, seven days a week.

source: resources

Record 31

topic: Clinical review

statement: The healthdirect breast augmentation page was last reviewed in November 2025 and its content is developed and reviewed under healthdirect's published content process.

source: what-is

references

Record 1

title: Breast Implants — Australian Journal of General Practice

url: https://www1.racgp.org.au/ajgp/2021/july/breast-implants-a-guide-for-general-practice

Record 2

title: Breastfeeding After Breast Augmentation Surgery: A Scoping Review

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11059175/

Record 3

title: The Psychological Impact of Aesthetic Surgery: A Mini-Review

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9687813/

Record 4

title: Advantages and Disadvantages of Breast Augmentation (Cureus)

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11492812/

Record 5

title: Social Media Influence on Body Image and Cosmetic Surgery Considerations (Cureus)

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11350482/

Record 6

title: Complications of reconstructive and aesthetic breast surgery (UpToDate)

url: https://www.uptodate.com/contents/complications-of-reconstructive-and-aesthetic-breast-surgery

Record 7

title: Breast Implants: Common Questions and Answers (American Family Physician)

url: https://www.aafp.org/pubs/afp/issues/2021/1100/p500.html

Record 8

title: Breast implants and anaplastic large cell lymphoma (TGA)

url: https://www.tga.gov.au/safety/safety-monitoring-and-information/safety-alerts/breast-implants-and-anaplastic-large-cell-lymphoma

Record 9

title: Current status of breast implant products in Australia (TGA)

url: https://www.tga.gov.au/safety/safety-monitoring-and-information/safety-alerts/current-status-breast-implant-

products-australia

Record 10

title: Five questions to ask your health professional before you get a medical implant (TGA)

url: https://www.tga.gov.au/products/medical-devices/specific-types-medical-devices/five-questions-ask-your-health-professional-you-get-medical-implant

Record 11

title: Breast implant safety patient information sheet (Agency for Clinical Innovation)

url: https://aci.health.nsw.gov.au/__data/assets/pdf_file/0007/935719/ACI-Breast-implant-safety-patient-information-sheet.pdf

Record 12

title: Patient information sheet: cosmetic surgery (Australian Government)

url: https://www.health.gov.au/resources/publications/patient-information-sheet-cosmetic-surgery

Record 13

title: Breast augmentation (Australian Society of Plastic Surgeons)

url: https://plasticsurgery.org.au/procedures/surgical-procedures/breast-augmentation-implants/

Record 14

title: AHPRA register of medical practitioners

url: https://www.ahpra.gov.au/registration/registers-of-practitioners.aspx

Record 15

title: FAQs — cosmetic surgery: requirement for GP referral (Medical Board of Australia)

url: https://www.medicalboard.gov.au/Codes-Guidelines-Policies/FAQ/FAQ-Cosmetic-surgery-Requirement-for-GP-referral.aspx

relatedPages

Record 1

title: Breast reduction

url: https://www.healthdirect.gov.au/procedures/surgery/breast-reduction

Record 2

title: Breast uplift

url: https://www.healthdirect.gov.au/procedures/surgery/breast-uplift

Record 3

title: Breast reconstruction with an implant or tissue expander

url: https://www.healthdirect.gov.au/procedures/surgery/breast-reconstruction-with-an-implant-or-tissue-expander

Record 4

title: Breast reconstruction with abdominal tissue flap

url: https://www.healthdirect.gov.au/procedures/surgery/breast-reconstruction-with-abdominal-tissue-flap

Record 5

title: Breast reconstruction with latissimus dorsi flap

url: https://www.healthdirect.gov.au/procedures/surgery/breast-reconstruction-with-latissimus-dorsi-flap

Record 6

title: Guide to cosmetic surgery

url: https://www.healthdirect.gov.au/procedures/surgery/guide-to-cosmetic-surgery

Record 7

title: Preparing for surgery

url: https://www.healthdirect.gov.au/procedures/surgery/preparing-for-surgery

Record 8

title: Mammography

url: https://www.healthdirect.gov.au/procedures/health-screenings/mammography

Research dataset: medlineplus-breast-enlargement

MedlinePlus Medical Encyclopedia (U.S. National Library of Medicine) — https://medlineplus.gov/

generated: 2026-10-20

source: MedlinePlus Medical Encyclopedia (U.S. National Library of Medicine)

sourceUrl: https://medlineplus.gov/

about: MedlinePlus patient information related to breast enlargement: breast augmentation surgery, implant fills and placement, anaesthesia and incision routes, risks including capsular contracture, breast implant-associated lymphoma and reported systemic symptoms, pre-operative preparation, recovery, wound care and the wider plastic and cosmetic surgery topic page.

categories

Record 1

id: augmentation

name: Breast augmentation surgery

description: The MedlinePlus encyclopedia article on the operation itself: what it is, how implants are placed, the anaesthetic and incision options, why it is performed, and the full risk list.

Record 2

id: recovery

name: Recovery, discharge and wound care

description: Patient instructions for the days and weeks after cosmetic breast surgery, plus general guidance for looking after a surgical incision that has been left open.

Record 3

id: related-procedures

name: Related breast procedures

description: Encyclopedia articles on other cosmetic breast operations that MedlinePlus indexes alongside augmentation.

Record 4

id: visual-resources

name: Videos and image series

description: Illustrated step-by-step series and the anatomy video that MedlinePlus attaches to its breast procedure articles.

Record 5

id: health-topic

name: Plastic and cosmetic surgery topic page

description: The MedlinePlus health topic hub that gathers the breast articles, image series, videos, statistics, clinical-trial searches and patient handouts.

documents

Record 1

id: breast-augmentation-surgery

title: Breast augmentation surgery

category: augmentation

kind: Medical Encyclopedia article

url: https://medlineplus.gov/ency/article/002984.htm

updated: 2025-02-17

summary: The MedlinePlus overview of breast augmentation: enlarging or reshaping the breasts by placing an implant behind the breast tissue or under the chest muscle fascia. The article covers the two implant fills (sterile saline and silicone), the outpatient or hospital setting, general versus local anaesthetic, the four usual incision routes, how implant and technique choices affect pain, appearance, implant integrity and future mammograms, the reasons for surgery, the risks (including capsular contracture, implant rupture, breast implant-associated lymphoma and reported systemic symptoms), what to arrange before the operation, what happens afterwards, and the expected outcome.

keyPoints

- Augmentation is done by placing an implant behind the breast tissue or beneath the chest muscle, and implants are sacs filled with either sterile salt water or silicone.
- The operation is performed at an outpatient surgery clinic or in a hospital, most often under general anaesthetic, with local anaesthetic as an alternative.
- Four incision routes are described: the natural fold under the breast (the most common), the armpit using an endoscope, the edge of the areola, and, for saline implants only, a route near the navel through which the empty implant is tunnelled and then filled.
- An areola-edge incision may bring more difficulty with breastfeeding and more loss of nipple sensation, and may carry a higher risk of capsular contracture.
- Implant type, implant size and the chosen operation influence post-operative pain, the appearance achieved, the chance of later breakage or leakage, and the extra views future mammograms will need.
- Reasons for the procedure include increasing breast size, changing breast shape and correcting a congenital deformity; patients are told to expect improvement rather than perfection.
- Risks shared with any operation are listed as reactions to medicines, breathing problems, bleeding, blood clots and infection.
- Breast-specific risks include difficulty breastfeeding, numbness around the nipple or surrounding skin, small or thickened raised scars, uneven nipple position, differences in size or shape between the two breasts, implant breakage or leakage, visible rippling, infection requiring implant removal, the need for more breast surgery and puncture of the lung.
- A capsule of scar tissue normally forms around a new implant; when it thickens it can change the breast's shape, harden the breast tissue or cause pain.
- A rare immune-system cancer, breast implant-associated lymphoma, has been reported with some implants, presenting as a mass or swelling around the implant with breast pain and treated by removing the implant and the tissue around it.
- Some people report systemic symptoms after implants — grouped under breast implant illness, which the page says is not yet established as a recognised illness — including fatigue, memory and concentration problems, joint and muscle pain, anxiety, hair loss, depression, rash, autoimmune disease, inflammation and weight change; some improve after removal and some do not.
- Before surgery patients should ask about a screening mammogram, disclose any pregnancy possibility and every medicine or supplement they take, review diabetes, heart disease and other conditions, stop smoking (which slows healing and raises clot risk), prepare the home and arrange transport and help for one to two days.
- During the week before surgery, blood thinners such as aspirin, ibuprofen, naproxen and vitamin E are commonly paused on the surgeon's advice, and fasting and morning-medicine instructions come from the

surgical team.

- Afterwards the breasts are wrapped in bulky gauze or a surgical bra, drainage tubes are removed within about three days, and massaging from around day five may be suggested to reduce capsular hardening after checking with the provider.
- Outcomes are usually very good, scars are permanent and most visible during the first year before fading, a supportive bra may be worn for a few months, and later tests may be advised to confirm the implants are intact.

Record 2

id: cosmetic-breast-surgery-discharge

title: Cosmetic breast surgery - discharge

category: recovery

kind: Patient instructions

url: https://medlineplus.gov/ency/patientinstructions/000273.htm

updated: 2025-02-17

summary: Home-care instructions after cosmetic breast surgery — augmentation, lift or reduction. It covers what happens in hospital, what to expect once home, activity and driving limits, dressing and wound care, pain medicines and general self-care, and the symptoms that should prompt a call to the surgical team.

keyPoints

- Surgery is done under general or local anaesthetic and takes an hour or more; patients wake with a gauze dressing or surgical bra and may have drainage tubes in place.
- Most people go home the same day, though some stay one to two nights in hospital.
- Pain, bruising and swelling are normal and settle over days to weeks, and reduced sensation in the breast skin or nipple may return gradually.
- Short walks are encouraged early to support circulation; returning to work or other activities commonly takes 7 to 14 days or longer.
- Heavy lifting, strenuous exercise and overstretching the arms are avoided for 3 to 6 weeks, and driving should wait at least two weeks and until drowsiness-inducing pain medicines have stopped.
- Drainage tubes are removed at a visit within a few days and stitches within two weeks; surgical glue dissolves on its own and does not need removing.
- Dressings stay in place as instructed, incisions are kept clean, dry and covered, and are checked daily for redness, pain or drainage.
- Once dressings are no longer needed, a soft wireless supportive bra is worn day and night for 2 to 4 weeks; bathing is limited to showering after drains come out, with baths, hot tubs and swimming deferred until stitches and drains are out and the surgeon agrees.
- Scars may take several months to more than a year to fade, and sunblock of SPF 30 or higher is advised whenever the scars are exposed.
- Smoking is not permitted because it slows healing and raises the risk of complications and infection; alcohol is avoided while taking narcotic pain medicines, and aspirin, ibuprofen and other medicines or supplements need the surgeon's approval.
- The surgeon should be contacted for increasing pain, redness, swelling, yellow or green drainage, bleeding or bruising at the incision, medicine side effects, a temperature of 100°F (38°C) or higher, numbness, loss of motion, or sudden swelling of the breast.

Record 3

id: surgical-wound-care-open

title: Surgical wound care - open

category: recovery

kind: Patient instructions

url: https://medlineplus.gov/ency/patientinstructions/000040.htm

updated: 2026-01-01

summary: General MedlinePlus instructions for looking after a surgical incision that has broken open and is being left to heal rather than re-closed with stitches. It explains healing from the inside out, hand hygiene before dressing changes, removing and replacing dressings, cleaning and irrigating the wound, disposal of used supplies, and the signs that mean a call to the provider. The breast augmentation article links this page for wound care after breast surgery.

keyPoints

- A wound left open heals from the inside out, and the dressing absorbs drainage so the surface skin does not close before the tissue underneath has filled in.
- Hands should be washed before every dressing change — an alcohol cleanser, or soap and water for 15 to 30 seconds including under the nails, with jewellery removed.
- The skin around the wound is cleaned with saline or mild soapy water; alcohol, peroxide, iodine and antibacterial cleansers are avoided because they can damage wound tissue and slow healing.
- Where irrigation is advised, a syringe of saline or soapy water is held about 2.5 to 15 cm from the wound to wash away discharge, then the area is patted dry.
- No lotion, cream or herbal remedy goes on or near the wound without the provider's agreement; dressings are used once only and soiled material is bagged before disposal.
- Contact the provider for more redness, pain, swelling or bleeding, a wound that looks larger, deeper, dried out or dark, thicker or foul-smelling discharge, or a temperature of 100.5°F (38°C) or higher.

Record 4

id: breast-lift

title: Breast lift

category: related-procedures

kind: Medical Encyclopedia article

url: https://medlineplus.gov/ency/article/007402.htm

summary: MedlinePlus Medical Encyclopedia article on breast lift (mastopexy), listed by MedlinePlus among the cosmetic breast procedures alongside augmentation and reduction, and referenced from the cosmetic breast surgery discharge instructions.

Record 5

id: breast-reduction

title: Breast reduction

category: related-procedures

kind: Medical Encyclopedia article

url: https://medlineplus.gov/ency/article/007405.htm

summary: MedlinePlus Medical Encyclopedia article on breast reduction (reduction mammoplasty), indexed on the Plastic and Cosmetic Surgery topic page and referenced from the cosmetic breast surgery discharge instructions as a related procedure.

Record 6

id: breast-augmentation-series

title: Breast augmentation - series

category: visual-resources

kind: Medical Encyclopedia image series

url: https://medlineplus.gov/ency/presentations/100205_1.htm

summary: Illustrated image series walking through breast augmentation, one of three breast procedure series (with lift and reduction) published in the MedlinePlus encyclopedia and linked from the breast augmentation article page.

Record 7

id: breast-lift-series

title: Breast lift (mastopexy) - series

category: visual-resources

kind: Medical Encyclopedia image series

url: https://medlineplus.gov/ency/presentations/100188_1.htm

summary: Illustrated image series covering breast lift (mastopexy), including the incision patterns used, published in the MedlinePlus encyclopedia.

Record 8

id: breast-reduction-series

title: Breast reduction (mammoplasty) - series

category: visual-resources

kind: Medical Encyclopedia image series

url: https://medlineplus.gov/ency/presentations/100189_1.htm

summary: Illustrated image series covering breast reduction (mammoplasty) and its indications, published in the MedlinePlus encyclopedia.

Record 9

id: breast-lift-video

title: Breast lift (anatomy video)

category: visual-resources

kind: Anatomy video

url: https://medlineplus.gov/ency/anatomyvideos/000017.htm

summary: MedlinePlus anatomy video explaining the breast lift procedure, embedded and recommended from the breast augmentation article page.

Record 10

id: plastic-cosmetic-surgery-topic

title: Plastic and Cosmetic Surgery

category: health-topic

kind: Health topic page

url: https://medlineplus.gov/plasticandcosmeticsurgery.html

updated: 2024-12-17

summary: The MedlinePlus health topic hub for plastic and cosmetic surgery. It summarises the body areas people most often have reshaped — including increasing, reducing or reshaping sagging breasts — and links out to the encyclopedia articles and image series, the breast lift video, ASPS plastic surgery statistics, ClinicalTrials.gov searches for breast implants and plastic surgery, and patient handouts including the breast

augmentation article in this library.

keyPoints

- Breast surgery appears among the most commonly requested cosmetic changes: enlarging, reducing or reshaping sagging breasts.
- The page indexes the breast augmentation, lift and reduction encyclopedia articles, their image series and the breast lift anatomy video.
- It links to FDA breast implant information, American Society of Plastic Surgeons statistics and ClinicalTrials.gov searches for breast implants and plastic surgery procedures.
- English and Spanish versions exist for the topic page and most of the linked resources.

facts

Record 1

topic: What breast augmentation is

statement: MedlinePlus describes breast augmentation as a procedure to enlarge the breasts or change their shape, carried out by placing an implant behind the breast tissue or under the muscle fascia or chest muscle.

source: breast-augmentation-surgery

Record 2

topic: Implant fills

statement: An implant is a sac filled with either sterile salt water (saline) or a material called silicone.

source: breast-augmentation-surgery

Record 3

topic: Setting and anaesthetic

statement: The operation is done at an outpatient surgery clinic or in a hospital, with most women receiving general anaesthesia while local anaesthesia — staying awake with the breast area numbed — is an alternative.

source: breast-augmentation-surgery

Record 4

topic: Incision routes

statement: Four approaches are described: a cut in the natural fold under the breast (the most common technique), a cut under the arm often using an endoscope (leaving no scar on the breast but possibly one under the arm), a cut around the areola edge, and, for saline implants, a cut near the navel through which the implant is moved up and filled.

source: breast-augmentation-surgery

Record 5

topic: Areola-edge trade-offs

statement: The periareolar route may bring more problems with breastfeeding and loss of sensation around the nipple, and may carry a higher risk of capsular contracture.

source: breast-augmentation-surgery

Record 6

topic: What changes the result

statement: The type of implant, its size and the type of surgery affect post-operative pain, the appearance of the breast, the risk of the implant breaking or leaking later, and the additional views future mammograms will

require.

source: breast-augmentation-surgery

Record 7

topic: Why it is performed

statement: The procedure is carried out to increase breast size, to change breast shape, or to correct a congenital deformity, with patients advised to discuss realistic expectations — improvement rather than perfection.

source: breast-augmentation-surgery

Record 8

topic: General surgical risks

statement: Risks applying to anaesthesia and surgery in general include reactions to medicines, breathing problems, bleeding, blood clots and infection.

source: breast-augmentation-surgery

Record 9

topic: Breast surgery risks

statement: Listed breast risks include difficulty breastfeeding, loss of feeling in the nipple area or surrounding skin, small or thickened raised scars, uneven nipple position, differences in size or shape between the breasts, breaking or leakage of the implant, visible rippling, removal of the implant because of infection, the need for further breast surgery and puncture of the lung.

source: breast-augmentation-surgery

Record 10

topic: Capsular contracture

statement: A capsule of scar tissue normally forms around a new implant to hold it in place; if this capsule thickens and grows it can change the breast's shape, harden breast tissue or cause pain.

source: breast-augmentation-surgery

Record 11

topic: Breast implant-associated lymphoma

statement: A rare lymphoma of the immune system has been reported with some implant types, presenting as a mass or swelling around the implant together with breast pain, and treated by removing the implant and the tissue around it; research into other cancer links is ongoing.

source: breast-augmentation-surgery

Record 12

topic: Breast implant illness

statement: Some people report systemic symptoms after receiving implants — fatigue, memory and concentration problems, joint and muscle pain, anxiety, hair loss, depression, rash, autoimmune disease, inflammation and weight gain or loss — described as breast implant illness, which the page says does not yet have enough evidence to be a recognised illness; some report improvement after implant removal and others do not.

source: breast-augmentation-surgery

Record 13

topic: Overall safety statement

statement: Despite these rare risks, MedlinePlus states that breast implants are considered safe.

source: breast-augmentation-surgery

Record 14

topic: Screening before surgery

statement: Patients are advised to ask whether a screening mammogram is needed given their age and breast cancer risk, arranged early enough that any further imaging or biopsy cannot delay the planned operation.

source: breast-augmentation-surgery

Record 15

topic: What to tell the surgeon

statement: The surgeon or nurse should be told about any pregnancy or possibility of pregnancy and about every medicine, drug, supplement or herb being taken, including items bought without a prescription.

source: breast-augmentation-surgery

Record 16

topic: Medical conditions and smoking

statement: Diabetes, heart disease and other conditions may require review by the provider who treats them, and smoking should be cut down or stopped because it slows healing and raises the risk of blood clots — surgery may be postponed while smoking continues.

source: breast-augmentation-surgery

Record 17

topic: Blood thinners before surgery

statement: During the week before surgery, medicines that stop blood clotting — including over-the-counter aspirin, ibuprofen, naproxen and vitamin E as well as many prescription medicines — are commonly stopped temporarily on the surgeon's advice, with instructions on which medicines to still take on the day of surgery.

source: breast-augmentation-surgery

Record 18

topic: Day-of-surgery instructions

statement: On the day of surgery patients follow the rules given about when to stop eating and drinking, take only the prescribed medicines with a small sip of water, and arrive at the hospital at the requested time.

source: breast-augmentation-surgery

Record 19

topic: Immediately after surgery

statement: Most patients go home once the anaesthetic has worn off and they can walk, drink water and use the bathroom safely; the breasts are wrapped in a bulky gauze dressing or a surgical bra, and drainage tubes, if placed, are removed within 3 days.

source: breast-augmentation-surgery

Record 20

topic: Massage after surgery

statement: Massaging the breasts from about 5 days after surgery may be recommended to reduce hardening of the capsule around the implant, but should only begin after checking with the provider.

source: breast-augmentation-surgery

Record 21

topic: Outcome and follow-up

statement: Outcomes from breast surgery are usually very good, pain and skin symptoms from the surgery generally settle, a special supportive bra may be worn for a few months, scars are permanent but typically fade after being most visible in the first year, and future tests may be recommended to confirm the implants have not ruptured.

source: breast-augmentation-surgery

Record 22

topic: Activity after discharge

statement: Home instructions after cosmetic breast surgery allow short walks early, put return to work or other activity at 7 to 14 days or longer, and prohibit heavy lifting, strenuous exercise and overstretching the arms for 3 to 6 weeks, with driving delayed at least 2 weeks.

source: cosmetic-breast-surgery-discharge

Record 23

topic: Signs to call the surgeon

statement: The surgical team should be contacted for increasing pain, redness, swelling, yellow or green drainage, bleeding or bruising at the incision, medicine side effects, a fever of 100°F (38°C) or higher, numbness or loss of motion, or sudden swelling of the breast.

source: cosmetic-breast-surgery-discharge

Record 24

topic: Scar care after discharge

statement: Incision scars may take several months to more than a year to fade, and protecting them with sunblock of SPF 30 or higher is advised whenever they are exposed to the sun.

source: cosmetic-breast-surgery-discharge

Record 25

topic: Healing an open incision

statement: Where an incision has broken open and is not re-closed, healing proceeds from the inside out and a dressing absorbs drainage so the surface does not close over before the wound underneath has filled in.

source: surgical-wound-care-open

Record 26

topic: Cleansers to avoid

statement: Saline or mild soapy water is used to clean around the wound, while skin cleansers, alcohol, peroxide, iodine and antibacterial soaps are avoided because they can damage wound tissue and slow healing.

source: surgical-wound-care-open

Record 27

topic: Wound infection warnings

statement: A provider should be contacted if redness, pain, swelling or bleeding increases, the wound looks larger, deeper, dried out or dark, discharge becomes thicker or smells of pus, or temperature reaches 100.5°F (38°C).

source: surgical-wound-care-open

Record 28

topic: Scope of the topic page

statement: The MedlinePlus Plastic and Cosmetic Surgery topic page lists increasing, reducing and reshaping sagging breasts among the most common cosmetic requests and indexes the breast augmentation, lift and reduction articles, image series and video.

source: plastic-cosmetic-surgery-topic

Research dataset: nhs-breast-enlargement

National Health Service (NHS, UK) — https://www.nhs.uk/

generated: 2026-10-20

source: National Health Service (NHS, UK)

sourceUrl: https://www.nhs.uk/

about: NHS patient information related to breast enlargement (implants), cosmetic procedure decision-making, choosing a practitioner, NHS funding, surgery abroad and reporting problems.

categories

Record 1

id: breast-enlargement

name: Breast enlargement (implants)

description: The NHS page on breast enlargement: what the operation involves, costs, implant choices, longevity, recovery, complications, BIA-ALCL, PIP implants, the implant registry and breast screening.

Record 2

id: deciding

name: Deciding whether to go ahead

description: NHS advice on working out whether a cosmetic procedure is right for you, and the steps to take before committing to any procedure.

Record 3

id: choosing-practitioner

name: Choosing who will do the procedure

description: How to check a surgeon or practitioner is properly qualified, registered and insured, and what to ask at a consultation.

Record 4

id: nhs-availability

name: Cosmetic surgery on the NHS

description: When the NHS may fund cosmetic procedures such as breast enlargement, and how that differs from reconstructive surgery.

Record 5

id: abroad

name: Cosmetic surgery abroad

description: NHS advice on weighing savings against risks when considering cosmetic surgery in another country, including aftercare and flying after surgery.

documents

Record 1

id: breast-enlargement

title: Breast enlargement (implants)

category: breast-enlargement

kind: Patient information page

url: https://www.nhs.uk/tests-and-treatments/cosmetic-procedures/cosmetic-surgery/breast-enlargement/

updated: 2023-09-21

summary: The NHS overview of breast enlargement (also called a boob job or breast augmentation). Implants are inserted to increase size, change shape or make breasts more even. The page covers funding, UK costs, choosing a surgeon and implants, how long implants last, the operation itself, recovery, complications, BIA-ALCL, breast implant illness, PIP implants, the implant registry, breast screening and how to report problems.

keyPoints

- Breast enlargement is major surgery; results are not guaranteed and there are risks to weigh up.
- It is usually paid for privately. The NHS may fund it in some circumstances, for example very uneven breasts or no breasts, depending on local rules.
- In the UK, breast implant surgery costs around £3,500 to £8,000, which does not usually include consultations or follow-up care.
- Two implant types are offered: silicone (most common in the UK) and saline. Placement is either behind the breast tissue or behind the chest muscle.
- Implants are not lifetime devices; some women need further surgery after about 10 years.
- The operation is done under general anaesthetic and takes between 60 and 90 minutes; most people go home the same day or stay overnight.
- Recovery takes a few weeks: one or two weeks off work, no driving for at least a week, and no heavy lifting or strenuous exercise for at least a month.
- Reported problems include capsular contracture, rupture, rippling, rotation, nipple sensation changes and reduced breast milk while breastfeeding.
- BIA-ALCL is an uncommon immune-system cell cancer associated with implants; breast implant illness is a term for symptoms some women report after surgery.
- PIP implants were banned in the UK in 2010; a ruptured implant should be removed as soon as possible.
- The Breast and Cosmetic Implant Registry (BCIR), set up in 2016, allows implants to be traced if a safety concern arises.
- Breast screening still matters after implants; mammograms are safe and can be performed differently so as much breast tissue as possible is seen.
- Check any hospital or clinic in England is registered with the CQC, and that the surgeon is on the GMC specialist register with a licence to practise; always meet the surgeon before the procedure.
- The MHRA publishes a Cosmetic Breast Augmentation Patient Risk Awareness Tool to help discuss implant risks and keep a record of treatment.

Record 2

id: right-for-me

title: Is a cosmetic procedure right for me?

category: deciding

kind: Advice page

url: https://www.nhs.uk/tests-and-treatments/cosmetic-procedures/advice/cosmetic-procedure-right-for-me/

updated: 2023-05-09

summary: NHS advice on working out whether a cosmetic procedure is a sound decision: questions to ask yourself, how to research, speaking to an expert, taking time, avoiding pressure and considering mental health.

keyPoints

- Ask what you want to change and why, how long you have thought about it, what triggered it now, and whether there are other ways to achieve the same change.
- Ask whether you are expecting the procedure to change your life as well as your appearance, and whether you want it for yourself or to please someone else.
- Consider all costs, including aftercare, correcting problems and future procedures needed to maintain the look.
- You should be given a cooling-off period after your consultation to decide.
- Avoid time-limited offers, extra treatments you had not considered, group treatment events involving alcohol, and mobile services operating in private homes or hotels.
- Tell the person treating you for a mental health condition, and your cosmetic practitioner, about any mental health problems past or present.
- Free NHS talking therapies are available for anxiety and depression if appearance concerns are affecting your wellbeing.

Record 3

id: before-you-have

title: Before you have a cosmetic procedure

category: deciding

kind: Advice page

url: https://www.nhs.uk/tests-and-treatments/cosmetic-procedures/advice/before-you-have-a-cosmetic-procedure/

updated: 2023-05-22

summary: NHS checklist for people considering a cosmetic procedure, surgical (such as breast implants) or non-surgical (such as dermal fillers or Botox): be clear about why, speak to an expert, research, understand costs, choose a qualified practitioner, ask questions, be cautious with social media and take time.

keyPoints

- All cosmetic procedures carry some risks, and complications can be serious if they are not done correctly.
- A GP or mental health professional can be a good first conversation, alongside a consultation with the practitioner.
- Understand the full costs including aftercare and any future procedures needed to maintain the result.
- Choose a practitioner who is trained, experienced and has the right insurance to correct anything that goes wrong.
- Social media influencers are usually paid to advertise services, so research anything found there with extra care.
- You should be offered a cooling-off period after any consultation, and you can walk away at any point.

Record 4

id: choosing-practitioner

title: Choosing who will do your cosmetic procedure

category: choosing-practitioner

kind: Advice page

url: https://www.nhs.uk/tests-and-treatments/cosmetic-procedures/advice/choosing-who-will-do-your-procedure/

updated: 2023-06-23

summary: NHS guidance on selecting a practitioner with the right training, skills and insurance: what to ask at a consultation, what the practitioner should explain, checks for cosmetic surgery in England (CQC clinic

registration, GMC medical register, RCS certification, BAAPS/BAPRAS membership) and registers for non-surgical practitioners.

keyPoints

- Always book a consultation with the person who will do the procedure before deciding.
- Ask how many procedures they have done, their qualifications, common complications, aftercare, what to do if something goes wrong, and the full cost including any further treatment.
- The practitioner should explain how the procedure is done, how long it takes, whether anaesthetic is needed, expected pain, recovery time, risks, how long results last and what to expect afterwards.
- Do not pay for a procedure until you have had a consultation.
- In England, all independent hospitals and clinics providing cosmetic surgery must be registered with the Care Quality Commission (CQC), which publishes quality ratings.
- Only registered doctors can perform cosmetic surgery in England; check the GMC medical register, and note the Royal College of Surgeons certifies cosmetic surgeons against a national standard.
- For non-surgical procedures, look for practitioners on PSA-accredited voluntary registers such as JCCP or Save Face.
- Avoid group treatments, treatment vouchers from discount sites, mobile services in homes or hotels, and practitioners who only advertise on social media.
- Dermal filler complications can be serious, including infection, nerve damage and blindness, so avoid practitioners with only short training courses.

Record 5

id: cosmetic-surgery-nhs

title: Is cosmetic surgery available on the NHS?

category: nhs-availability

kind: Advice page

url: https://www.nhs.uk/tests-and-treatments/cosmetic-procedures/advice/cosmetic-procedures-on-the-nhs/

updated: 2023-04-17

summary: Cosmetic surgery is not routinely provided on the NHS, though it may occasionally be funded for psychological or other health reasons. Breast enlargement may be funded for very uneven breasts or no breasts causing significant psychological distress. Reconstructive plastic surgery, such as breast rebuilding after mastectomy or cleft lip and palate repair, is different and often available.

keyPoints

- Most people who want cosmetic surgery pay for it privately.
- NHS breast enlargement funding depends on local commissioning rules; your GP can explain the rules in your area.
- Other examples where the NHS may pay include nose reshaping for breathing problems, ear correction, and breast reduction causing backache.
- Male breast reduction may be funded for long-standing gynaecomastia that has not responded to other treatments and causes considerable pain or distress.
- Reconstructive surgery aims to repair or reconstruct missing or damaged tissue after illness, accident or birth defect, and is often available on the NHS.

Record 6

id: surgery-abroad

title: Cosmetic surgery abroad

category: abroad

kind: Advice page

url: https://www.nhs.uk/tests-and-treatments/cosmetic-procedures/advice/cosmetic-surgery-abroad/

updated: 2022-12-14

summary: NHS advice on cosmetic surgery abroad: lower prices must be weighed against risks, follow-up care may be missing or of a different standard, holiday packages need caution, and aftercare, insurance and flying plans should be sorted out in advance.

keyPoints

- Overseas clinics may not provide follow-up treatment, or not to the same standard as the UK, and may have no UK professional you can see if problems arise.
- Be cautious of websites selling cosmetic surgery as part of a holiday: meet the surgeon, and do not pay for a hospital you have never seen or a surgeon you have not met.
- Recovering straight after a holiday is unrealistic; rest is required, with no alcohol, sunbathing, sightseeing or swimming.
- Find out what happens if there are complications, where follow-ups will be, how easily you could travel back for more surgery, and how much it would cost.
- Standards and qualifications abroad may differ, so find out how surgeons and clinics are regulated in the chosen country.
- You should have two consultations with the treating surgeon before surgery, giving a cooling-off period; they should explain the procedure, its limitations, alternatives and risks, and show before-and-after pictures.
- Travel insurance normally does not cover things going wrong with surgery; check clinic insurance, travel cover and how you would get home in an emergency.
- Do not fly for 5 to 7 days after procedures such as breast surgery, as air travel plus major surgery increases the risk of a life-threatening blood clot.

facts

Record 1

topic: Funding

statement: Breast enlargement is usually paid for privately. The NHS may fund it in some circumstances, for example very uneven breasts or no breasts, and it often depends on the rules in your local area; your GP can explain them.

source: breast-enlargement

Record 2

topic: NHS funding exceptions

statement: Cosmetic surgery is not routinely provided on the NHS, but may be provided for psychological or other health reasons, such as breast enlargement causing significant psychological distress.

source: cosmetic-surgery-nhs

Record 3

topic: Cost in the UK

statement: Breast implant surgery costs around £3,500 to £8,000 in the UK. This does not usually include consultations or follow-up care, and future follow-up surgery is also payable.

source: breast-enlargement

Record 4

topic: What the operation does

statement: Implants are inserted to increase breast size, change their shape or make breasts more even. The procedure is often known as a boob job or breast augmentation.

source: breast-enlargement

Record 5

topic: Implant types

statement: There are two types of implant: silicone and saline. Silicone is the most common in the UK — less likely to wrinkle and feels more natural, but can spread into the breast and cause lumps. Saline is more likely to fold, rupture or deflate over time, but leaked saline is safely absorbed by the body.

source: breast-enlargement

Record 6

topic: Implant placement

statement: Implants are placed either behind the breast tissue or behind the chest muscle, and size, shape and placement are discussed with the surgeon before the operation.

source: breast-enlargement

Record 7

topic: Implant longevity

statement: Breast implants do not last a lifetime and will probably need replacing at some point. Some women need further surgery after about 10 years because of implant problems or because the breasts have changed around the implants.

source: breast-enlargement

Record 8

topic: The operation

statement: Breast implant surgery is usually done under general anaesthetic. An incision is made next to or below the breast, the implant is positioned, and the incision is stitched and dressed. The operation takes between 60 and 90 minutes, with same-day discharge or an overnight stay.

source: breast-enlargement

Record 9

topic: Recovery

statement: Take one or two weeks off work, do not drive for at least one week, and avoid heavy lifting or strenuous exercise for at least a month. Stitches come out after one to two weeks, most normal activity resumes after about six weeks, and breasts settle over a few months. Some surgeons recommend a sports bra 24 hours a day for up to three months.

source: breast-enlargement

Record 10

topic: Sunbathing and flying

statement: It is safe to sunbathe and fly if you have breast implants, once recovered. Separately, the NHS advises not flying for 5 to 7 days after breast surgery because of blood clot risk.

source: breast-enlargement

Record 11

topic: Common implant problems

statement: Breast implants can cause thick scarring, capsular contracture (the breast feeling hard as scar tissue

shrinks around the implant), rupture, creases or folds, rotation, rippling, nipple nerve changes that may be temporary or permanent, and slightly less breast milk while breastfeeding.

source: breast-enlargement

Record 12

topic: General surgical risks

statement: As with any operation there is a small risk of bleeding and clots (blood clots can be life threatening), infection treated with antibiotics, and allergic reaction to medicines or products used during surgery such as antibiotics or latex.

source: breast-enlargement

Record 13

topic: Rupture detection

statement: A ruptured implant may cause small tender lumps (siliconomas) visible only on breast scans, and the implant will need to be removed. Ultrasound or MRI scans can check whether implants have ruptured.

source: breast-enlargement

Record 14

topic: BIA-ALCL

statement: There is an association between breast implants and an uncommon immune system cell cancer called breast implant associated anaplastic large cell lymphoma (BIA-ALCL), which develops in the scar tissue around implants in a very small number of people.

source: breast-enlargement

Record 15

topic: Breast implant illness

statement: Some women report symptoms after breast implant surgery such as tiredness, anxiety and joint pain, sometimes referred to as breast implant illness.

source: breast-enlargement

Record 16

topic: PIP implants

statement: PIP (Poly Implant Prostheses) implants were banned in the UK in 2010 after they were found to contain unapproved silicone gel and to be more likely to split. Anyone with PIP implants should get medical advice, and a ruptured implant should be removed as soon as possible.

source: breast-enlargement

Record 17

topic: Implant registry

statement: The Breast and Cosmetic Implant Registry (BCIR) was set up in 2016 to record the details of anyone who has breast implants for any reason, so they can be traced if there is a safety concern about a specific implant type.

source: breast-enlargement

Record 18

topic: Screening after implants

statement: You can still get breast cancer after implants and should continue regular breast screening. Mammograms are safe and do not rupture implants; because X-rays cannot pass through implants, the

mammogram may be done differently so as much breast tissue as possible can be seen.

source: breast-enlargement

Record 19

topic: Reporting problems

statement: Problems with breast implants can be reported through the government's Yellow Card Scheme. Concerns about care can go to the Care Quality Commission, and concerns about a doctor to the General Medical Council.

source: breast-enlargement

Record 20

topic: Choosing a clinic in England

statement: All independent clinics and hospitals providing cosmetic surgery in England must be registered with the Care Quality Commission (CQC), which publishes quality ratings online.

source: choosing-practitioner

Record 21

topic: Choosing a surgeon

statement: Check the surgeon is registered with the General Medical Council (GMC), listed on the specialist register and holding a licence to practise. Many are also members of BAPRAS or BAAPS, whose registers can be checked, and the Royal College of Surgeons certifies cosmetic surgeons against an agreed national standard.

source: choosing-practitioner

Record 22

topic: Questions for your surgeon

statement: The NHS suggests asking about qualifications and experience, professional association membership, how many breast enlargements they have performed, how many operations involved complications, the implant type, manufacturer and why it is used, the surgical technique and placement, expected results, follow-up if things go wrong, patient satisfaction rates and alternative options.

source: breast-enlargement

Record 23

topic: Risk awareness tool

statement: The Medicines and Healthcare products Regulatory Agency (MHRA) publishes a Cosmetic Breast Augmentation Patient Risk Awareness Tool (PDF) that patients can use to discuss potential implant risks with their surgeon and keep a record of their treatment.

source: breast-enlargement

Record 24

topic: Further information

statement: The NHS points to patient information from the British Association of Aesthetic Plastic Surgeons (BAAPS), the British Association of Plastic, Reconstructive and Aesthetic Surgeons (BAPRAS) and the Royal College of Surgeons for more on breast augmentation and cosmetic surgery.

source: breast-enlargement

Record 25

topic: Time to decide

statement: You should be given a cooling-off period after your consultation. The NHS warns against time-limited offers, extra treatments pushed at the clinic, group events involving alcohol, and mobile services operating in private homes or hotels.

source: right-for-me

Record 26

topic: Payment

statement: Do not pay for a procedure until you have had a consultation to make sure it is right for you.

source: choosing-practitioner

Record 27

topic: Social media advertising

statement: Be cautious about procedures or clinics found on social media; influencers are usually paid to advertise, and practitioners who only advertise on social media should be avoided.

source: before-you-have

Record 28

topic: Surgery abroad

statement: Cosmetic surgery abroad can cost less, but follow-up care may be missing or of a different standard, travel insurance normally does not cover surgical complications, and you should have two consultations with the surgeon before surgery. Do not fly for 5 to 7 days after breast surgery.

source: surgery-abroad

Research dataset: pmc-breast-enlargement

PubMed Central (PMC), U.S. National Library of Medicine — https://pmc.ncbi.nlm.nih.gov/

generated: 2026-10-19

source: PubMed Central (PMC), U.S. National Library of Medicine

sourceUrl: https://pmc.ncbi.nlm.nih.gov/

query: "breast enlargement"

total: 1874

yearRange

- 1841
- 2027

withAbstract: 1559

journals: 741

articles

Record 1

pmcid: PMC13579683

pmid: 42751413

doi: 10.1016/j.ctro.2026.101284

title: Acute breast edema during 5-fraction ultra-hypofractionated radiotherapy after breast-conserving surgery: A cone-beam CT based evaluation.

authors

- Zhang Y
- Zhang W
- Chang C
- Qi S
- Song Y
- Fang H
- Tang Y
- Chen S
- Zhai Y
- Zhang W
- Huan F
- Hong T
- Li X
- Men K
- Yi J
- Jing H

journal: Clinical and translational radiation oncology

journalAbbr: Clin Transl Radiat Oncol

year: 2027

pubdate: 2027 Jan

volume: 62

issue:

pages: 101284

abstract: BACKGROUND: With the growing adoption of 5-fraction ultra-hypofractionated radiotherapy (uHFRT) following breast-conserving surgery (BCS), in-treatment breast edema has been increasingly observed. Marked edema may impair target dose coverage and even necessitate radiotherapy interruption. This study aimed to investigate the incidence, evolution and risk factors of this acute adverse event. METHODS: Patients treated with BCS were enrolled in a prospective phase II trial of uHFRT. Patients received whole-breast irradiation (WBI) at 26 Gy in 5 fractions, with or without simultaneous integrated tumor-bed boost (SIB) of 30 Gy, delivered daily or on alternate days. Cone-beam CT (CBCT) was acquired prior to each fraction. Acute breast edema was defined as outward expansion of the breast contour exceeding 1 mm relative to the planning CT. Clinical and target volume parameters were analyzed to identify risk factors. RESULTS: Among 221 patients (224 breasts), edema occurred in 83 breasts (37.1%), with 96.4% developing after the first fraction. The prevalence of edema was 35.7%, 36.6%, 37.1%, and 35.3% at the 2nd to 5th pretreatment CBCT scans, respectively. Edema peaked after the first fraction in 42 swelling breasts (50.6%). Contour expansion was >3-5 mm in 19.6%, >5-8 mm in 7.1%, >8-10 mm in 1.8%, and > 10 mm in 0.9% of all breasts. Multivariate analysis indentified younger age and greater breast thickness as significant factors associated with the occurrence of in-treatment breast edema. Furthermore, breast thickness was positively associated with edema severity. CONCLUSIONS: The incidence of acute in-treatment breast edema during uHFRT after BCS was 37.1%, with onset immediately after the first fraction in 96.4% of affected cases. Contour expansion >5 mm was observed in 9.8% of the entire cohort. Given the short treatment course and high single-fraction dose, early-onset edema may impair treatment efficacy. Close surveillance and timely intervention are warranted for high-risk patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13579683/

Record 2

pmcid: PMC13095568

pmid: 42022318

doi: 10.3389/fonc.2026.1769477

title: A longitudinal observational study of male referrals and adherence to imaging guidelines: a single centre experience in 148 patients.

authors

- Pestrin O
- Agilinko J
- Basharat SH
- Ali T
- Wahab T

journal: Frontiers in oncology

journalAbbr: Front Oncol

year: 2026

pubdate: 2026

volume: 16

issue:

pages: 1769477

abstract: INTRODUCTION: Male breast cancer represents a rare clinical diagnosis, accounting for less than 1%

of all breast cancers. The vast majority of male breast referrals are attributed to benign pathologies, most commonly gynaecomastia. Despite the low incidence of malignancy, variability in clinical acumen and a low threshold for concern frequently result in excessive imaging. To mitigate this, the Association of Breast Surgery (ABS) has issued national guidelines to standardise assessment and reduce unnecessary radiological exposure. This study aimed to evaluate male breast referrals to a high-volume UK breast unit over one year and assess adherence to ABS imaging recommendations. METHODS: We conducted a retrospective analysis of all male patients referred to the symptomatic breast clinic at King George Hospital between January and December 2022. Clinical presentation, P-scores, radiological imaging (M and U scores), histopathological outcomes, and medication history were extracted from hospital records and multidisciplinary systems. Imaging pathways were critically appraised against 2021 ABS guidance, stratifying patients by age (<25 and ≥25 years). RESULTS: A total of 148 male patients were assessed, with a mean age of 49.6 years (SD ± 22.3). The most frequent presentation was a unilateral breast lump (62.8%). Gynaecomastia was diagnosed in 97 patients (65.5%), with proton pump inhibitors implicated in over a quarter of these cases. Imaging was performed in 141 patients (95.3%), with 52.7% of all imaging deemed non-concordant with ABS guidance. Only 29.7% underwent imaging entirely in keeping with national recommendations. Eight patients (5.4%) were diagnosed with breast cancer, all over the age of 25. Notably, only one malignancy arose in a patient with a benign clinical examination (P1), flagged by indeterminate mammographic findings (M3). Advanced cross-sectional imaging (CT/MRI) was performed in 2% of the cohort. CONCLUSION: Despite clear national guidelines, our findings highlight a significant trend towards over-investigation in male breast referrals. The predominance of benign disease particularly drug-induced gynaecomastia underscores the need for judicious imaging. Adherence to ABS criteria must be reinforced, and closer collaboration between breast surgeons and radiologists is essential to optimise diagnostic yield, reduce patient anxiety, and conserve healthcare resources in the evaluation of the male breast.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13095568/

Record 3

pmcid: PMC13009130

pmid: 41162798

doi: 10.1007/s00266-025-05196-x

title: A New Clinical Classification Method for Gynecomastia Based on Chest Appearance and MR/CT Images.

authors

- Gu J
- Hu H
- Ying J
- Xu Q
- Lu L
- Yu X
- Zhuang N
- Zhou J
- Jiang H
- Wang H

journal: Aesthetic plastic surgery

journalAbbr: Aesthetic Plast Surg

year: 2026

pubdate: 2026 Mar

volume: 50

issue: 5

pages: 1797-1810

abstract: BACKGROUND: Classification theories for gynecomastia often conflate breast ptosis degree with subcutaneous soft tissue composition, leading to further confusion in determining appropriate surgical regions and techniques. To address this, we propose a new clinical classification method for gynecomastia. METHODS: We first divided each side of male chest into six distinct regions, then separated the concept of grade and type in gynecomastia classification. Five grades were defined based on chest appearance and breast ptosis (skin redundancy). Grade 1-4 Patients underwent surgery in region 1-4 accordingly, with region 5-6 addressed if necessary. Grade 5 patients received treatment in all six regions. Two types with six subtypes were defined based on the proportion and distribution of subcutaneous glandular tissue shown in MR/CT images. Type 1 (A-C) patients typically required liposuction alone, with pull-through added if necessary, while Type 2 (A/B) patients generally required additional pull-through. For Type 2C patients, mammotome-assisted resection was used to remove dense, enlarged glands. RESULTS: Two hundred and ninety-seven gynecomastia patients were included in this study. There were 32, 183, 45, 12, and 5 patients in grade 1-5, respectively, along with 20 patients with asymmetric gynecomastia. A total of 132 patients underwent preoperative MR/CT imaging, with 85 classified as Type 1, 39 as Type 2, and 8 as asymmetric cases. CONCLUSION: We propose a new gynecomastia classification method based on chest appearance and MR/CT images, distinguishing grade from type. Grade determines the surgical region, while type guides the choice of surgical technique. This approach is objective and clinically meaningful. LEVEL OF EVIDENCE IV: This journal requires that authors assign a level of evidence to each article. For a full description of these Evidence-Based Medicine ratings, please refer to the Table of Contents or the online Instructions to Authors www.springer.com/00266 .

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13009130/

Record 4

pmcid: PMC13521103

pmid: 42662570

doi: 10.1016/j.jpra.2026.07.037

title: A novel technique for pectoral enhancement performed during excisional gynaecomastia procedures.

authors

- Glynn N
- Currie R
- Hodgins N

journal: JPRAS open

journalAbbr: JPRAS Open

year: 2026

pubdate: 2026 Sep

volume: 51

issue:

pages: 750-754

abstract: The presence of gynaecomastia can result in significant psychological distress. Surgical intervention should aim to remove glandular tissue whilst ensuring an aesthetically pleasing result. We present a case of excisional gynaecomastia correction with simultaneous enhancement of the pectoral contour with a modified dermal flap. This method utilises otherwise redundant autologous tissue without an additional donor site. We report an aesthetically pleasing result and discuss other modalities of pectoral enhancement currently described in the literature.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13521103/

Record 5

pmcid: PMC13054666

pmid: 41953099

doi: 10.1002/ccr3.72227

title: A Rare Case of Adenoid Cystic Carcinoma in the Male Breast: Clinical Presentation and Histopathological Evaluation.

authors

- Tayebi N
- Jahanbin B
- Nozheh A

journal: Clinical case reports

journalAbbr: Clin Case Rep

year: 2026

pubdate: 2026 Apr

volume: 14

issue: 4

pages: e72227

abstract: Adenoid cystic carcinoma (ACC) of the male breast is an exceptionally rare neoplasm with a distinct clinical behavior. We report a case of a 40-year-old man who presented with ACC of the right breast. The patient was initially treated with surgical excision; however, a local recurrence was identified just 4 months postoperatively. This rapid recurrence underscores the aggressive potential of ACC in this unique demographic. The case highlights the critical importance of rigorous postoperative surveillance and a more extensive consideration of adjuvant therapies to mitigate recurrence risk. Further research is essential to establish optimal management protocols for this rare disease.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13054666/

Record 6

pmcid: PMC13657606

pmid: 42866748

doi: 10.1097/MS9.0000000000005608

title: A rare case report: atypical presentation of Klinefelter syndrome in a 24-year-old male at suburban tertiary care hospital in Nepal.

authors

- Bastola B
- Bajracharya B
- Yadav AK
- Tamrakar R

journal: Annals of medicine and surgery (2012)

journalAbbr: Ann Med Surg (Lond)

year: 2026

pubdate: 2026 Oct

volume: 88

issue: 10

pages: 6807-6811

abstract: Klinefelter syndrome (KS) is the most common chromosomal condition affecting males, usually caused by the presence of an extra X chromosome (47, XXY). However, it often remains undiagnosed until adulthood. We report a unique case of a 24-year-old man whose primary concern was persistent nausea and vomiting, an atypical presenting complaint of Klinefelter syndrome. Further evaluation revealed typical features of KS, including infertility despite normal sexual desire, gradual enlargement of the breasts, tall stature, sparse facial and body hair, small testes, and learning difficulties. An uncommon finding in the patient was premature loss of dentition. Hormonal tests showed low testosterone levels with elevated gonadotropins and estradiol, consistent with primary hypogonadism, and karyotype analysis confirmed the diagnosis. This case highlights the wide spectrum of clinical features associated with Klinefelter syndrome, including both typical and atypical manifestations. This report emphasizes the need for increased awareness of such unusual presentations, for early diagnosis, and a multidisciplinary approach to management that can improve physical health, psychological well-being, and overall quality of life.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13657606/

Record 7

pmcid: PMC13400338

pmid: 42502786

doi: 10.1016/j.radcr.2026.06.151

title: A rare presentation of male breast schwannoma.

authors

- Naidoo D
- Molina MER
- Gryaznov A
- Lewis E
- Jacob J
- Gruen D

journal: Radiology case reports

journalAbbr: Radiol Case Rep

year: 2026

pubdate: 2026 Oct

volume: 21

issue: 10

pages: 4550-4554

abstract: Breast schwannomas are rare benign tumors originating from Schwann cells of peripheral nerves. They are exceedingly uncommon in the male breast and are often misdiagnosed as gynecomastia, particularly in patients on prolactin-elevating medications. We report a case of a 40-year-old male with a longstanding palpable mass in the upper outer quadrant of the left chest wall. The patient had a history of schizoaffective disorder and was on antipsychotic therapy, initially leading to the clinical presumption of gynecomastia. Diagnostic mammography revealed a well-circumscribed, noncalcified mass adjacent to the pectoralis muscle. Ultrasound demonstrated a heterogeneously hypoechoic lesion with cystic changes, classified as BI-RADS 4. Ultrasound-guided core biopsy revealed a schwannoma with S100 positivity which was confirmed on final surgical pathology after excision. This case underscores the importance of maintaining a broad differential for male breast masses and highlights the limitations of imaging alone in distinguishing rare benign neoplasms from malignancy. Biopsy remains essential for diagnosis, particularly in patients with confounding clinical risk factors.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13400338/

Record 8

pmcid: PMC13101556

pmid: 42028376

doi: 10.1016/j.jpra.2026.03.025

title: A retrospective population-based analysis of 19,640 pediatric and young adult males with gynecomastia.

authors

- Hilewitz D
- Tsion GB
- Ad-El D
- Meshulam-Derazon S
- Kalish E
- Olshinka A

journal: JPRAS open

journalAbbr: JPRAS Open

year: 2026

pubdate: 2026 May

volume: 49

issue:

pages: 595-606

abstract: Gynecomastia is the most common breast condition in pediatric and young adult males, yet large-scale studies evaluating its etiologies, risk factors, and management remain scarce. In adolescents, gynecomastia is most frequently idiopathic or obesity-related, while secondary causes are rarer. Persistent presentations may cause psychosocial distress and lead to surgical correction, though the optimal diagnostic approach remains controversial. We conducted a retrospective, population-based study of 19,640 males aged 10-25 years with documented gynecomastia diagnosis, extracting demographic and clinical variables including age, body mass index (BMI) Z-scores, family history, comorbidities, medication exposure, and surgical status. Reporting was in accordance with the STROBE guidelines. Most patients were idiopathic (74.3%), followed by medication-related (23.1%) and pathology-associated (3.4%). For patients with pathology-associated cases compared with other etiologies, the mean age and mean BMI Z-score were higher, and surgeries were more frequent (all $p < 0.001$). The prevalence of pathology increased with age, whereas obesity-related and familial cases predominated in younger children. Morbid obesity was significantly associated with higher surgical rates (5.2% vs. 2.1% in non-obese, $p < 0.001$). Independent associations with surgical intervention included older age (odds ratio (OR 1.20), higher BMI Z-score (OR 1.30), ≥2 affected siblings (OR 8.35), and testicular tumors (OR 14.06). Model discrimination (area under the curve = 0.973) was excellent. In conclusion, gynecomastia in younger boys is primarily idiopathic or obesity-related, while older adolescents are more often diagnosed with secondary causes. Age- and risk-tailored diagnostic strategies, including early screening for obesity, family history, and medication exposure may optimize management and guide surgical decision-making.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13101556/

Record 9

pmcid: PMC13447672

pmid:

doi: 10.1177/1742271X261454870

title: Abstracts from the 56th Annual Scientific Meeting of the British Medical Ultrasound Society, 9th – 11th December 2025, Harrogate Convention Centre, United Kingdom.

authors

journal: Ultrasound: Journal of the British Medical Ultrasound Society

journalAbbr: Ultrasound

year: 2026

pubdate: 2026 Aug

volume: 34

issue: 3

pages: 226-68

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13447672/

Record 10

pmcid: PMC12838742

pmid: 41595266

doi: 10.3390/cancers18020348

title: Achieving Pregnancy After Early Hormone Receptor-Positive Breast Cancer: Recent Evidence and Clinical Considerations.

authors

- Ronan KE
- Walshe JM

journal: Cancers

journalAbbr: Cancers (Basel)

year: 2026

pubdate: 2026 Jan 22

volume: 18

issue: 2

pages:

abstract: An increasing number of young women with hormone receptor-positive (HR+) early breast cancer desire pregnancy after treatment. Prolonged adjuvant endocrine therapy, concerns regarding recurrence risk, and treatment-related fertility decline have historically complicated reproductive decision-making in this population. This narrative review synthesizes current evidence on pregnancy after early HR+ breast cancer, with particular emphasis on prospective data from the POSITIVE trial. We examine the safety of temporary endocrine therapy interruption, the impact of assisted reproductive technologies (ART) in achieving pregnancy, breastfeeding feasibility and impact, hormonal predictors of fertility, pregnancy outcomes and considerations for special populations, including BRCA mutation carriers. Retrospective studies have suggested no adverse survival impact associated with pregnancy after breast cancer. The POSITIVE trial provides prospective evidence that temporary interruption of endocrine therapy to attempt pregnancy does not increase short-term recurrence risk in selected patients. Approximately three-quarters of participants achieved pregnancy. Fertility preservation and ART were commonly used and were not associated with worse short-term oncologic outcomes. Biomarkers such as anti-Müllerian hormone offer supportive but imperfect prediction of fertility potential. Breastfeeding was feasible for many women and did not adversely affect breast cancer outcomes. Available data among BRCA mutation carriers are reassuring but largely observational. Current evidence supports the safety and feasibility of pregnancy after early HR+ breast cancer in carefully selected patients. However, longer follow-up, inclusion of higher-risk populations, and evaluation of newer therapies are needed. Individualized, multidisciplinary counselling remains central to informed decision-making.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12838742/

Record 11

pmcid: PMC12783840

pmid: 41507446

doi: 10.1038/s41386-025-02281-2

title: ACNP 64th Annual Meeting: Poster Abstracts P584-P872.

authors

journal: Neuropsychopharmacology : official publication of the American College of Neuropsychopharmacology

journalAbbr: Neuropsychopharmacology

year: 2026

pubdate: 2026 Jan

volume: 51

issue: Suppl 1

pages: 410-571

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12783840/

Record 12

pmcid: PMC12941451

pmid: 41753176

doi: 10.3390/jcm15041487

title: Advances and Challenges in Obstetric Intensive Care Medicine.

authors

- Braga A
- De Melo HK
- Paiva G
- Rodrigues GM
- Callado GY
- Araujo Júnior E
- Amim-Junior J
- de Rezende-Filho J
- Granese R

journal: Journal of clinical medicine

journalAbbr: J Clin Med

year: 2026

pubdate: 2026 Feb 13

volume: 15

issue: 4

pages:

abstract: Obstetric critical care encompasses the management of pregnant and postpartum women with life-threatening conditions, requiring integration of intensive care principles with pregnancy-specific physiological,

ethical, and organizational considerations. Although pregnancy is a physiological state, profound maternal adaptations may mask early signs of clinical deterioration, allowing rapid progression to a critical illness condition. This review provides a comprehensive overview of the foundations of obstetric intensive care, addressing maternal-fetal physiology, recognition of severity, organ support strategies, and contemporary models of care. Key aspects discussed include cardiovascular, respiratory, renal, and hematological adaptations of pregnancy; principles of airway management and mechanical ventilation; hemodynamic support; transfusion strategies guided by viscoelastic testing; renal replacement therapy; extracorporeal support, including extracorporeal membrane oxygenation and cardiopulmonary bypass; and the safe use of diagnostic imaging involving ionizing radiation. The role of point-of-care ultrasonography, structured early warning systems, and advanced monitoring in early detection and management of clinical deterioration is emphasized. Special attention is given to maternal-fetal interactions, fetal monitoring in the intensive care unit (ICU), and complex decision-making regarding timing and mode of delivery. The review also highlights the importance of multidisciplinary and multiprofessional collaboration, ethical challenges inherent to dual-patient care, and emerging strategies to expand access to specialized care, including tele-ICU models and artificial intelligence-assisted surveillance. Across all scenarios, maternal stabilization remains the primary determinant of fetal outcome. A structured approach grounded in maternal-fetal physiology and ethical principles is essential to reduce preventable maternal and perinatal morbidity and mortality in high-complexity settings.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12941451/

Record 13

pmcid: PMC13518669

pmid: 42645664

doi: 10.1186/s13244-026-02383-5

title: AI performance varies considerably across mammography devices: a multi-site and multi-vendor retrospective study.

authors

- Blum M
- Morant R
- Eichenberger A
- Geissler A
- Subelack J
- Vogel J
- Ehlig D

journal: Insights into imaging

journalAbbr: Insights Imaging

year: 2026

pubdate: 2026 Aug 26

volume: 17

issue: 1

pages:

abstract: OBJECTIVES: To evaluate an artificial intelligence (AI) algorithm on three different mammography devices and assess screening performance using general and device-specific thresholds. MATERIALS AND METHODS: This retrospective study analyzed 22,673 AI-assessed mammographies of women aged 50-69 collected in 2022-2023 from three mammography devices across six screening sites within a Swiss Mammography Screening Program. 115 screen-detected and 15 1-year interval breast cancers were included in the analyses. A commercially available AI algorithm assessed each mammography with a numeric case score. Optimal thresholds were determined across all devices and for each device separately using the Youden-Index.

Screening performance was assessed using sensitivity, specificity, balanced accuracy, and cancer detection rate (CDR). RESULTS: Standard double-reading had a CDR of 5.07 (95% CI: 4.19-6.09). Using AI with a general threshold achieved a CDR of 4.81 (95% CI: 3.95-5.80), which was enhanced using device-specific thresholds to 4.90 (95% CI: 4.03-5.89). Workload for radiologists in consensus conferences was lowered by nearly one-third when using device-specific thresholds compared to a general threshold ($p < 0.001$). Nonetheless, AI screening performance for one of the mammography devices was poor even with device-specific thresholds. CONCLUSION: AI-based performance varied considerably among the three mammography devices, emphasizing the need for device-specific thresholds or even the withdrawal of AI use on certain devices. Awareness of different case distribution among radiologists should be raised to mitigate the risk of interpretation biases and reduced diagnostic accuracy. Lastly, training datasets must be representative not only of the target population but also of the different screening devices used in practice. KEY POINTS: Question AI performance may vary across mammography devices, raising uncertainty about device-independent thresholds. Findings AI scores differed considerably across devices, indicating the need for device-specific thresholds, but performance on one device remained poor. Critical relevance The performance of an AI algorithm in mammography screening depends on adequately defined decision thresholds, and radiologists must be aware that AI scores can vary across mammography devices to avoid interpretation biases and to maintain diagnostic accuracy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13518669/

Record 14

pmcid: PMC12952879

pmid: 41912224

doi: 10.5826/dpc.1601a5872

title: An Atypical Neonatal Genital Crisis.

authors

- El Bouhmadi A
- El Fatoiki FZ
- Hali F
- Chiheb S

journal: Dermatology practical & conceptual

journalAbbr: Dermatol Pract Concept

year: 2026

pubdate: 2026 Jan 30

volume: 16

issue: 1

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12952879/

Record 15

pmcid: PMC13483729

pmid: 42260423

doi: 10.1186/s12884-026-09419-0

title: Antenatal care utilisation among pregnant adolescents in urban Ghana: a qualitative study of individual, family and health system influences.

authors

- Opoku-Mensah K
- Blankson NA
- Ayimbila EA
- Glozah F

journal: BMC pregnancy and childbirth

journalAbbr: BMC Pregnancy Childbirth

year: 2026

pubdate: 2026 Jun 8

volume: 26

issue: 1

pages:

abstract: BACKGROUND: Adolescent pregnancy remains an important public health concern in Ghana and other low- and middle-income settings. Pregnant adolescents often initiate antenatal care later than recommended or attend irregularly, increasing the risk of adverse maternal and newborn outcomes. While determinants of antenatal care use have been widely studied, less is known about how individual, family, and health system factors interact to shape adolescents' care seeking experiences. This study explored influences on antenatal care utilisation among pregnant adolescents in an urban municipality in southern Ghana. METHODS: A qualitative descriptive study was conducted in the La Nkwantanang Madina Municipality in the Greater Accra Region of Ghana. Data were collected through in-depth interviews, focus group discussions, and key informant interviews with pregnant adolescents, caregivers, midwives, and community members. Interviews were conducted in English or Twi, audio recorded, transcribed verbatim, and analysed thematically. RESULTS: Forty-two participants were included: 15 pregnant adolescents, 6 caregivers, 5 midwives, and 16 community members. Three interrelated themes shaped antenatal care utilisation: individual influences, family influences, and health system influences. Delayed pregnancy recognition, stigma, and financial constraints contributed to late initiation of care. Family responses, including support or disapproval, influenced attendance patterns. Health system factors such as provider attitudes, waiting time, privacy concerns, and out of pocket costs also shaped utilisation, although supportive care experiences encouraged continued attendance. CONCLUSION: Antenatal care utilisation among pregnant adolescents is shaped by interacting individual, family, and health system influences. Strengthening adolescent responsive services, supportive family engagement, and reducing structural barriers may improve timely and consistent antenatal attendance.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13483729/

Record 16

pmcid: PMC13232851

pmid: 42234610

doi: 10.1371/journal.pone.0330399

title: Antenatal education for labour and postpartum pain: A scoping review of content, delivery approaches, evidence gaps, and lived experiences.

authors

- Sloyan E
- Leddy E
- Clark C
- Dufour S
- Harper R
- Dunford A

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journal: PloS one

journalAbbr: PLoS One

year: 2026

pubdate: 2026

volume: 21

issue: 6

pages: e0330399

abstract: BACKGROUND: Pain during labour and the postpartum period is a complex and multidimensional experience. Antenatal education programmes aim to prepare individuals for childbirth and early parenthood; however, the extent to which these programmes address labour and postpartum pain management, and how women experience this education, remains unclear. This scoping review aimed to map the content, delivery characteristics, and evidence gaps of antenatal education programmes addressing labour and postpartum pain, including women's lived experiences. METHODS: This review was conducted in accordance with PRISMA-ScR and Joanna Briggs Institute guidelines. The protocol was registered with the Open Science Framework (6597j). Twelve electronic databases were systematically searched in November 2025. Quantitative, qualitative, and mixed-methods primary studies examining antenatal education programmes with a focus on labour or postpartum pain were included. A narrative synthesis was undertaken to map intervention content, delivery approaches, and pain-related outcomes and experiences. RESULTS: A total of 5,959 records were identified from the search strategy. A total of 17 articles met the eligibility and inclusion criteria, including seven randomised controlled trials, seven quasi-experimental studies, one pre-post study, and two qualitative studies. The content and structure of antenatal education interventions between studies was heterogenous. Common themes included the distinction between "true and false labour pain" and breathing exercises. Qualitative findings highlighted women's perceived improvements in pain coping, confidence, sense of control, and use of non-pharmacological strategies during labour. CONCLUSION: Antenatal education programmes contain limited information on labour and postpartum pain management, with little consistency across interventions. While non-pharmacological strategies appear valuable in supporting coping and confidence during labour, pain mechanisms and postpartum pain remain under-addressed. Incorporating pain-focused education such as pain neuroscience principles may enhance antenatal education and support more effective pain management. Further research is required to develop and evaluate consistent, evidence-based antenatal education approaches that address both labour and postpartum pain.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13232851/

Record 17

pmcid: PMC13103885

pmid: 42018313

doi: 10.1001/jamapsychiatry.2026.0428

title: Antipsychotic Efficacy and Safety of LB-102 in the Treatment of Adults With Acute Schizophrenia: A Randomized Clinical Trial.

authors

- Eramo A
- Correll CU
- Walling DP
- Kakar R
- Bassani N
- Callahan L
- Lee BP

- Prensky Z
- Vaino AR
- Kane JM

journal: JAMA psychiatry

journalAbbr: JAMA Psychiatry

year: 2026

pubdate: 2026 Jul 1

volume: 83

issue: 7

pages: 682-693

abstract: IMPORTANCE: LB-102 (N-methyl amisulpride) is a novel benzamide under investigation for the treatment of schizophrenia. OBJECTIVE: To evaluate the efficacy and safety of LB-102 in acute schizophrenia. DESIGN, SETTING, AND PARTICIPANTS: This US-based, multicenter, double-blind, placebo-controlled, phase 2 randomized clinical trial (NOVA1) conducted from December 2023 to August 2024 comprised an inpatient screening period (≤14 days), 28-day inpatient treatment period, 5-day inpatient stabilization period, and outpatient safety follow-up visit approximately 2 weeks after the treatment period end. Eligible participants were adults (aged 18-55 years) with schizophrenia who required hospitalization or continued hospitalization for an acute exacerbation of psychotic symptoms and had a Positive and Negative Syndrome Scale (PANSS) total score of 80 to 120, PANSS Positive Symptoms subscale item score of 4 or greater on 2 or more key items, and a Clinical Global Impressions-Severity of Illness scale (CGI-S) score of 4 or greater at screening and baseline. INTERVENTIONS: Participants were randomized (3:3:3:1) to oral once-daily placebo, LB-102 50 mg, 75 mg, or 100 mg. MAIN OUTCOMES AND MEASURES: The primary end point was change from baseline to week 4 in PANSS total score (Hochberg multiplicity correction for LB-102 50 mg and 75 mg vs placebo). Secondary efficacy end points included changes from baseline to week 4 in CGI-S score, CGI-S response, PANSS subscale scores, PANSS Marder factor scores, and 20% or greater PANSS response. Safety and tolerability end points included treatment-emergent adverse effects (TEAEs). RESULTS: A total of 359 participants (mean [SD] age, 39.1 [9.3] years; 290 male [80.8%]; mean baseline PANSS total score = approximately 94 across groups) were randomized and included in the safety and intention-to-treat populations (placebo, n = 108; 50 mg, n = 107; 75 mg, n = 108; 100 mg, n = 36). The trial met the primary end point: LB-102 50 mg and 75 mg, were statistically superior to placebo in change from baseline to week 4 in PANSS total score (mean [SE], placebo, -9.3 [1.08]; 50 mg, -14.3 [1.10], P < .001; Hedges g = 0.61; 75 mg, -14.0 [1.11], P = .002; Hedges g = 0.41); LB-102 100 mg, also showed significance (mean [SE], -16.1 [1.91]; nominal P = .002; Hedges g = 0.83). TEAEs were reported in 60 participants (56%) in the placebo group, 74 (69%) in the group receiving 50 mg, 62 (57%) in the group receiving 75 mg, and 27 (75%) in the group receiving 100 mg. Ten participants reported TEAEs leading to withdrawal (placebo, n = 2; 50 mg, n = 2; 75 mg, n = 3; 100 mg, n = 3) with 5 serious TEAEs (placebo, n = 2 [including 1 death]; each LB-102 arm, n = 1). CONCLUSIONS AND RELEVANCE: This randomized clinical trial provided rigorous evidence demonstrating the efficacy and safety of LB-102 for the treatment of adults with acute schizophrenia. TRIAL REGISTRATION: ClinicalTrials.gov Identifier: NCT06179108.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13103885/

Record 18

pmcid: PMC12939260

pmid: 41751407

doi: 10.3390/cimb48020143

title: Are Peripartum Changes in CCL2 Associated with Maternal Metabolic Status?

authors

- Obuchowska-Standyło A

- Kimber-Trojnar Ż
- Trojnar K
- Czuba M
- Leszczyńska-Gorzelak B

journal: Current issues in molecular biology

journalAbbr: Curr Issues Mol Biol

year: 2026

pubdate: 2026 Jan 28

volume: 48

issue: 2

pages:

abstract: C-C motif chemokine ligand 2 (CCL2) may reflect subtle metabolic-inflammatory changes in pregnancy. This study evaluated CCL2 concentrations and their peripartum changes in women with uncomplicated term pregnancies, focusing on associations with maternal metabolic status. Serum CCL2 was measured before delivery and 48 h postpartum; urinary CCL2 was assessed postpartum. Peripartum serum change (ΔCCL2) was calculated. BMI was recorded pre-pregnancy (or early pregnancy), at delivery, and 48 h postpartum; total BMI change (ΔBMI) was derived. Participants were stratified into two groups (ΔBMI > 1 kg/m 2 vs. ≤1 kg/m 2). Peripartum serum CCL2 changes differed significantly between ΔBMI groups. In the total cohort, CCL2 correlated with HbA1c and selected body composition indices, including fat tissue index, lean tissue index, and body cell mass. In women with ΔBMI > 1 kg/m 2 , additional associations were found with BMI, peripartum BMI change, HbA1c, ferritin, creatinine, and total body water. Among women with ΔBMI ≤ 1 kg/m 2 , significant relationships were observed with uric acid and triglycerides. Peripartum CCL2 dynamics appear to reflect maternal metabolic status, even in metabolically "normal" pregnancies, but these findings are exploratory and should be interpreted cautiously. CCL2 is a promising marker of subtle metabolic alterations in late pregnancy and the early postpartum period, but further validation is required before clinical application.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12939260/

Record 19

pmcid: PMC12892513

pmid: 41546057

doi: 10.1186/s12905-026-04259-8

title: Associations between social media use, body mass index, and body image among pregnant women.

authors

- Töre FK
- Ağralı C
- Nacar G

journal: BMC women's health

journalAbbr: BMC Womens Health

year: 2026

pubdate: 2026 Jan 16

volume: 26

issue: 1

pages: 91

abstract: BACKROUND: Body image during pregnancy is an important psychosocial factor that influences

maternal well-being. Emerging evidence suggests that both social media exposure and body mass index (BMI) may shape body image perceptions during this period. This study examined the associations between social media use, BMI, and body image in pregnant women. METHODS: This cross-sectional, descriptive, and correlational study was conducted with 272 pregnant women recruited through convenience sampling from the gynecology and obstetrics outpatient clinics of a state hospital in the Mediterranean region of Türkiye. Data were collected using a Personal Information Form, the Social Media Addiction Scale–Adult Form, and the Body Image in Pregnancy Scale. RESULTS: Correlation analysis showed that higher social media addiction scores were significantly associated with higher BIPS scores ($r = 0.366$, $p < 0.001$), whereas BMI was not significantly correlated ($r = 0.090$, $p = 0.140$) with body image Multiple linear regression analysis further demonstrated that satisfaction with body before pregnancy ($\beta = 0.137$, $p = 0.024$), satisfaction with body changes during pregnancy ($\beta = 0.205$, $p = 0.003$), emotions when away from social media ($\beta = 0.161$, $p = 0.016$), and social media addiction ($\beta = 0.347$, $p = 0.001$) were significantly associated with higher negative body image scores, while worry about postpartum body appearance was inversely associated with BIPS scores ($\beta = -0.155$, $p = 0.026$). The model explained 30.5% of the variance in body image, with social media addiction emerging as the strongest predictor. CONCLUSION: This study demonstrated that higher levels of social media use and addiction were associated with more negative body image perceptions among pregnant women. Health professionals may consider integrating counseling and educational interventions into prenatal care to promote healthier and more mindful social media use during pregnancy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12892513/

Record 20

pmcid: PMC13076940

pmid: 41988341

doi: 10.1093/jscr/rjag247

title: Asymptomatic breast implant rupture detected in pregnancy: a case report and a management pathway.

authors

- Nguyễn NM
- Võ TT
- Trần SB
- Nguyễn HM
- Phạm HT

journal: Journal of surgical case reports

journalAbbr: J Surg Case Rep

year: 2026

pubdate: 2026 Apr

volume: 2026

issue: 4

pages: rjag247

abstract: Breast implant rupture during pregnancy is rarely reported, and physiologic breast changes may obscure clinical detection. Management should prioritize maternal-fetal safety with rupture-related concerns. We report a gravida 3 woman with a 7-year-old textured silicone implant placed subpectorally in whom left intracapsular rupture was incidentally suspected during a breast ultrasound performed alongside routine antenatal assessment at 32 + 5 weeks' gestation. She remained asymptomatic, with no palpable abnormalities, inflammatory changes, or systemic signs. After multidisciplinary counseling, she was managed expectantly with scheduled clinical reviews and defined return precautions. Magnetic resonance imaging (MRI) was deferred during pregnancy because results were unlikely to change antepartum management. Postpartum MRI confirmed left implant rupture. Elective surgery and implant exchange seven months postpartum was uncomplicated. This case supports ultrasound-led, safety-netted observation until postpartum, and reinforces routine surveillance

and preconception implant checks for women planning pregnancy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13076940/

Record 21

pmcid: PMC13390014

pmid: 42487662

doi: 10.1002/ccr3.73193

title: Azoospermia, Atrophy, and Asymmetry: Unilateral Sertoli Cell Tumor as a Rare and Overlooked Cause of Male Infertility: A Case Report and Literature Review.

authors

- Ali RN
- Irfan W
- Irfan S
- Khan MA
- Bibi M
- Ali U
- Javaid MH
- Tague C

journal: Clinical case reports

journalAbbr: Clin Case Rep

year: 2026

pubdate: 2026 Jul

volume: 14

issue: 7

pages: e73193

abstract: Sertoli cell tumors are uncommon sex-cord stromal tumors making up < 2% of testicular tumors and have been reported to be associated with endocrine dysfunction that could compromise male fertility. Herein, we present a 34-year-old Pakistani male with primary infertility, azoospermia, enlargement of right testis, and contralateral severe testicular atrophy. Hormone analysis showed decreased testosterone level, high follicle-stimulating hormone, and high levels of estradiol with normal levels of serum tumor markers. Ultrasonography and computed tomography revealed a right intratesticular well-defined mass with severe testicular atrophy of the left testis. Right radical orchidectomy was done. Histopathology revealed benign NOS-type Sertoli cell tumor with positive staining for vimentin and calretinin. Non-tumorous testicular parenchyma showed severe impairment in spermatogenesis with Johnsen score 2-3. After the procedure, normalizing hormones were observed but azoospermia persisted. Our report underscores the occurrence of contralateral testicular atrophy with azoospermia due to a unilateral Sertoli cell tumor probably due to chronic estrogen elevation. Evaluation of patients with Sertoli cell tumors for endocrine disorders, fertility assessments, and discussion of fertility preservation measures including onco-testicular sperm extraction needs to be considered.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13390014/

Record 22

pmcid: PMC13264789

pmid: 42299268

doi: 10.21037/gs-2026-1-0133

title: Bilateral primary diffuse large B-cell lymphoma of the breast: diagnostic and surgical management-a case

report.

authors

- Hao Z
- Zheng M
- Zhang B

journal: Gland surgery

journalAbbr: Gland Surg

year: 2026

pubdate: 2026 May 31

volume: 15

issue: 5

pages: 148

abstract: BACKGROUND: Primary breast lymphoma (PBL) is a rare extranodal lymphoma of which diagnosis is challenging due to overlapping clinical and imaging features with breast carcinoma. While the existence of PBL is known, the diagnostic significance of specific sonographic patterns like the "grid-like echo" and the critical imperative for exhaustive systemic staging to differentiate primary from systemic lymphoma warrant emphasis. This case is unique as it demonstrates a diagnostic journey from a bilateral "grid-like echo" on ultrasound to a revised diagnosis of a systemic, high-grade lymphoma, highlighting key pitfalls and the necessity of comprehensive workup. CASE DESCRIPTION: A 38-year-old female presented with systemic symptoms. Bilateral breast ultrasound revealed Breast Imaging-Reporting and Data System (BI-RADS) 4 lesions with a distinctive "grid-like echo" pattern, well-defined hypoechoic masses, and rich vascularity. Core needle biopsy confirmed a high-grade B-cell lymphoma. However, comprehensive staging-including bone marrow biopsy, cytogenetics, and positron emission tomography-computed tomography (PET-CT)-revealed widespread disease, a t(8;14) translocation, MYC rearrangement, and multifocal extranodal involvement. The final diagnosis was revised to systemic Burkitt-like lymphoma with 11q aberration, Ann Arbor Stage IV B. The patient received systemic chemotherapy, resulting in symptomatic improvement and resolution of breast lesions on follow-up. CONCLUSIONS: While ultrasonography holds significant value in detecting and suggesting PBL, definitive diagnosis depends entirely on pathological evidence acquired through surgical biopsy. The cornerstone of treatment is systemic chemotherapy, and radical surgical resection does not improve patient survival. Therefore, the indispensable role of surgery lies in its diagnostic, not therapeutic, capacity. Enhancing awareness of PBL and establishing a multidisciplinary diagnostic model that integrates surgery, ultrasonography, pathology, and clinical practice is essential to avoid misdiagnosis and develop individualized treatment strategies.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13264789/

Record 23

pmcid: PMC13625182

pmid: 42819910

doi: 10.3389/fpsyg.2026.1960364

title: Bio-banding in football: a scoping review with evidence gap map.

authors

- Zhang J
- Bao R
- Wang L
- Zhuge S
- Pan P
- An D

- Chen J
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journal: Frontiers in psychology

journalAbbr: Front Psychol

year: 2026

pubdate: 2026

volume: 17

issue:

pages: 1960364

abstract: AIM: This study conducted the first scoping review with evidence gap map (EGM) on bio-banding in football, a field that has received increasing attention. Although previous studies have investigated technical and physical performance, including physiological outcomes, these findings have not yet been comprehensively reviewed. Hence, this review aimed to synthesize the existing evidence, identify research gaps, and inform future research priorities. METHODS: This scoping review followed the PRISMA Extension for Scoping Reviews (PRISMA-ScR) guidelines. The databases searched were Web of Science, Scopus, SPORTDiscus, and ScienceDirect, and the searches were conducted on 29 January 2026. Eligible studies were original articles published in English or Spanish that investigated bio-banding in youth football players aged 11-15 years, with no restriction on publication year. Data on study characteristics, participant information, and study outcomes were extracted and synthesized to develop the evidence gap map (EGM). RESULTS: A total of 47 records were identified, of which 20 met the selection criteria. Publications on bio-banding have increased markedly in recent years. Technical performance and psychological outcomes were the most frequently investigated themes, whereas no studies examined injury outcomes. The majority of studies involved male players and were conducted in Europe, highlighting the need for research involving female players, stakeholders, and more geographically diverse populations. CONCLUSION: Bio-banding is a promising strategy for talent identification and player development. Current evidence suggests that it may reduce maturity-related differences while supporting technical, tactical, and psychological development. However, important evidence gaps remain, particularly regarding physiological outcomes, female players, non-European populations, and diverse study designs. These findings provide practical guidance for coaches and practitioners and highlight priorities for future research.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13625182/

Record 24

pmcid: PMC12856948

pmid: 41626238

doi: 10.1093/jscr/rjag028

title: Bloody nipple discharge in young children: mammary duct ectasia as a benign, self-limiting condition.

authors

- Al Blooshi M
- Abdulrahman DD
- Saad WS
- Javaid U
- Gupta V

journal: Journal of surgical case reports

journalAbbr: J Surg Case Rep

year: 2026

pubdate: 2026 Jan

volume: 2026

issue: 1

pages: rjag028

abstract: Bloody nipple discharge in children is rare and often alarms families because of its malignant associations in adults. We describe two prepubertal boys (3 and 5 years) with unilateral, spontaneous, painless bloody nipple discharge without trauma, infection, systemic symptoms, or exogenous hormone exposure. Examinations showed only mild unilateral breast change, with no masses, skin or nipple abnormalities, or lymphadenopathy. Breast ultrasonography was the primary investigation: one child had retroareolar ductal dilatation with cystic change consistent with mammary duct ectasia; the other had normal breast architecture. When obtained, laboratory, microbiologic, and cytologic studies were unremarkable. Both patients were managed conservatively with caregiver counseling and structured clinical/sonographic follow-up. Discharge resolved spontaneously within 14 and 8 months, respectively, with complete clinical and imaging resolution and no recurrence. These cases support ultrasound-based assessment and expectant management when no concerning features are present.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12856948/

Record 25

pmcid: PMC13611596

pmid: 42784019

doi: 10.3390/reports9030294

title: Brachydactyly Type A1 Caused by an IHH Variant in a Patient with Disproportionate Short Stature: A Case Report.

authors

- García de Pablo I
- Ontoria Betancort MC
- Martínez Bugallo F
- Martín Pérez SE
- Martín Pérez IM

journal: Reports (MDPI)

journalAbbr: Reports (MDPI)

year: 2026

pubdate: 2026 Sep 1

volume: 9

issue: 3

pages:

abstract: Introduction and Clinical Significance: Skeletal dysplasias comprise a genetically heterogeneous group of disorders with substantial phenotypic overlap, often complicating diagnosis. Clinical exome sequencing (CES) can facilitate molecular diagnosis in children with unexplained disproportionate short stature. Case Presentation: An 8-year-old boy presented with severe short stature (-3.24 SDS), brachydactyly, relative macrocephaly, broad nasal bridge, and mild calf hypertrophy. Endocrine evaluation confirmed growth hormone deficiency (GHD). Following negative SHOX testing, CES identified a heterozygous likely pathogenic IHH variant (c.446G>A; p.Arg149His), establishing the diagnosis of brachydactyly type A1 (BDA1). Recombinant human growth hormone (rhGH), initiated for GHD, resulted in improved growth velocity and height SDS. Transient unilateral prepubertal gynecomastia developed during treatment and resolved after temporary rhGH withdrawal, with no recurrence following reinitiation. Conclusions: This case highlights the diagnostic value of CES in children with

disproportionate short stature after unrevealing targeted testing and illustrates that GHD may coexist with IHH - related skeletal dysplasia. An integrated genetic and endocrine evaluation can refine diagnosis, identify coexisting treatable endocrine disorders, and guide individualized management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13611596/

Record 26

pmcid: PMC12854579

pmid: 41414894

doi: 10.4103/idoj.idoj_588_24

title: Breast Lymphoma: Rare Mimicker of Breast Cancer.

authors

- Dharia AM
- Jangid KM
- Tambe SA
- Mahobia S

journal: Indian dermatology online journal

journalAbbr: Indian Dermatol Online J

year: 2026

pubdate: 2026 Jan 1

volume: 17

issue: 1

pages: 119-125

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12854579/

Record 27

pmcid: PMC13327674

pmid: 42394758

doi: 10.1159/000552263

title: Breast Mass as the Initial Presentation of B-Cell Acute Lymphoblastic Leukemia: A Case Report.

authors

- Zahreddin A
- Alabed A
- Al-Bitar A
- Hossein Morteza M
- Tellawi I
- Alsayaleh A

journal: Case reports in oncology

journalAbbr: Case Rep Oncol

year: 2026

pubdate: 2026 Jan-Dec

volume: 19

issue: 1

pages: 848-856

abstract: INTRODUCTION: Leukemic infiltration of the breast is an exceedingly rare clinical entity, often posing a significant diagnostic challenge as it can mimic common benign or malignant breast conditions. The presence of inflammatory features may further obscure the diagnosis, delaying appropriate intervention. CASE PRESENTATION: A 17-year-old female presented with a palpable right breast mass and axillary lymphadenopathy. Initial imaging revealed a 2.8 × 3.7 cm hypoechoic mass with indistinct margins and increased vascularity. Clinical findings were misinterpreted as an infectious or inflammatory process, particularly following excision of a reactive axillary lymph node. Her course was complicated by axillary vein thrombosis and a large pleural effusion. Subsequent clinical deterioration prompted a peripheral blood smear and bone marrow biopsy, which revealed B-cell acute lymphoblastic leukemia with 40% blasts, confirmed by immunohistochemistry (positive for LCA, CD20, PAX5, TdT). A breast biopsy performed after 1 month of chemotherapy revealed persistent extramedullary disease despite bone marrow remission. Treatment was escalated to hyper-CVAD, but the patient succumbed to complications, including neutropenic fever, pneumonia, and paraplegia. CONCLUSION: This case underscores the diagnostic consequences of leukemic breast infiltration. A high index of suspicion for hematological malignancy is essential when evaluating breast masses, particularly in young patients with atypical features, unexplained systemic symptoms, or a progressive course despite antibiotics. Fine-needle aspiration (FNA) coupled with immunophenotyping or flow cytometry is a rapid and effective diagnostic tool that can prevent unnecessary surgeries and ensure the timely initiation of systemic chemotherapy for managing this aggressive disease.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13327674/

Record 28

pmcid: PMC12909515

pmid: 41710015

doi: 10.3389/fped.2026.1759715

title: Breast masses in childhood: a single-center experience.

authors

- Temuroğlu A
- Aksoy GR
- Özşen M
- Gürpınar AN
- Sevinir BB

journal: Frontiers in pediatrics

journalAbbr: Front Pediatr

year: 2026

pubdate: 2026

volume: 14

issue:

pages: 1759715

abstract: OBJECTIVE: Pediatric breast masses are rare conditions. Although most of them are benign, they can cause concern in families. The present study aims to determine breast masses' clinical and pathological outcomes in childhood. MATERIAL AND METHOD: The records of patients who underwent further evaluations for breast masses between 2010 and 2023 at a single center were retrospectively reviewed. RESULTS: A total of 32 patients with breast tumors were included in the study. The median age of the patients was 16 years (1-18 years); 90.6% (n = 29) were female, and 9.4% (n = 3) were male. Most patients, 90.6% (n = 29), had a

painless, palpable mass. A family history of breast cancer was present in four patients. One patient had received chemotherapy for neuroblastoma and one for teratoma. The most common location was the upper outer quadrant in 35.5% of patients (n = 11). Bilateral mass involvement was present in five patients (15.6%). The mean tumor size was 32.64 ± 17.4 mm (range 9-80 mm). The mean tumor diameter was 24.6 ± 14.2 mm in patients who were followed without surgery and 39.2 ± 17.4 mm in those who underwent surgery (P = 0.017). A biopsy was performed in 53.1% (n = 17) of the patients, and surgery in 56.2% (n = 18). The most frequent pathology was fibroepithelial lesion and fibroadenomas 57.1% (n = 20). Malignant tumors (leiomyosarcoma and T cell lymphoma) were observed in 6.3% of the patients (n = 2) and borderline phyllodes tumors in 18.7% (n = 6). Recurrence was observed in 18.7% of the patients (n = 6) during the follow-up. CONCLUSION: In childhood, the most commonly encountered breast tumors are benign. However, careful monitoring is crucial due to the potential occurrence of malignant tumors. Further evaluations should be undertaken in patients with a history of malignancy or radiotherapy, masses larger than 5 cm, or masses with progressive growth.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12909515/

Record 29

pmcid: PMC13164038

pmid: 42121583

doi: 10.3390/healthcare14091140

title: Breast Reconstruction After Cancer: Historical Development, Modern Techniques, and Psychological Impact.

authors

- Tušak M
- Porčnik A
- Kneževič I
- Markovič-Božič J
- Tušak M
- Lapoša A

journal: Healthcare (Basel, Switzerland)

journalAbbr: Healthcare (Basel)

year: 2026

pubdate: 2026 Apr 24

volume: 14

issue: 9

pages:

abstract: Breast reconstruction represents an integral component of contemporary breast cancer management, with substantial impact on patients' psychological well-being, body image, and overall quality of life. Given the profound symbolic and personal significance of the breast, mastectomy-whether total or partial-extends beyond oncologic resection and may result in considerable aesthetic, functional, and psychosocial consequences. For this reason, reconstructive planning should be incorporated into the initial multidisciplinary treatment strategy while ensuring that oncologic safety and adjuvant therapies are never compromised. Breast reconstruction may be achieved using autologous tissue, implant-based techniques, or a combination of both approaches. Each method carries specific advantages, limitations, and potential complications and must be tailored to the individual patient's oncologic status, anatomy, and expectations. This article provides a historical overview of the evolution of breast cancer treatment and reconstructive techniques. It further examines the principles, benefits, and challenges associated with different reconstructive modalities, highlighting key considerations in clinical decision-making and long-term outcomes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13164038/

Record 30

pmcid: PMC13461050

pmid: 42583527

doi: 10.1097/GOX.0000000000008027

title: Breast Sculpting: An Anatomically Based Reconstruction During 408 Breast Explantations.

authors

- Maciel PJ
- Daronch OT
- Cherem AF
- Tamaro C
- Nassar F

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2026

pubdate: 2026 Aug

volume: 14

issue: 8

pages: e8027

abstract: BACKGROUND: Over time, there has been a significant rise in the number of patients who require and/or desire breast explantation. From implant complication issues to more contemporary reasons, such as breast implant disease, the causes are varied. METHODS: This retrospective study analyzed 204 patients (408 breasts) who underwent explantation between 2021 and 2025 using the "breast sculpting" technique. The approach incorporates strategies such as tissue recruitment, muscle repositioning, and reshaping of the breast footprint, with the objective of restoring contour and achieving optimal aesthetic balance. RESULTS: About 78.6% of the implants were in the retroglandular plane, 11.5% in the submuscular plane, and 9.8% in the dual plane. Two (0.98%) patients developed deep vein thrombosis, 1 (0.49%) patient experienced a minor pulmonary embolism, and 5 (1.22%) patients experienced unilateral breast hematomas. CONCLUSIONS: The breast sculpting technique promotes a tailored approach to breast tissue management. It facilitates multiple strategies for reconstructing breast shape following implant removal, with a particular emphasis on restoring natural breast contour-thereby differentiating it from previously described methods in the literature. This technique demonstrates a low risk of complications and does not preclude the use of lipofilling as a subsequent procedure.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13461050/

Record 31

pmcid: PMC13200334

pmid: 42178583

doi: 10.1186/s13058-026-02253-4

title: British society of breast radiology annual scientific meeting abstracts.

authors

journal: Breast cancer research : BCR

journalAbbr: Breast Cancer Res

year: 2026

pubdate: 2026 May 25

volume: 28

issue: Suppl 1

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13200334/

Record 32

pmcid: PMC13626914

pmid: 42823955

doi: 10.3389/fonc.2026.1883637

title: Case Report: Metachronous bilateral primary and secondary angiosarcoma of the breast.

authors

- Zhuang S
- Zhang J
- Chen M
- Lai Y
- Chen L

journal: Frontiers in oncology

journalAbbr: Front Oncol

year: 2026

pubdate: 2026

volume: 16

issue:

pages: 1883637

abstract: BACKGROUND: Breast angiosarcoma (AS) is an extremely rare and aggressive malignancy with no standardized treatment guidelines. We report an exceptional case of metachronous bilateral primary and secondary AS in a single patient, highlighting the diagnostic and therapeutic challenges posed by this disease. CASE PRESENTATION: A patient underwent surgical dissection for primary AS of the right breast. Seven months later, a primary AS was identified in the left breast. Four months after that, a secondary angiosarcoma also developed within the previously irradiated left chest wall and axilla. Postoperative pathology confirmed angiosarcoma in all resected specimens. Following multidisciplinary team discussion, the patient received simple mastectomy, adjuvant chemotherapy, and radiotherapy. Despite this multimodal approach, contrast-enhanced CT follow-up revealed rapid disease progression with distant metastases to the lungs, liver, spleen, bilateral adnexa, and bones. The patient ultimately died of respiratory failure secondary to severe pulmonary infection and atelectasis 22 months after the initial diagnosis. CONCLUSION: This extraordinarily rare case of sequential bilateral primary and secondary breast AS underscores the aggressive biological behavior of this malignancy and the limitations of current therapeutic strategies. It highlights the urgent need for identifying specific diagnostic biomarkers and novel therapeutic targets to enable early detection and improve the dismal prognosis associated with this disease.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13626914/

Record 33

pmcid: PMC13175789

pmid: 42147117

doi: 10.3389/fendo.2026.1799027

title: Case Report: rapid progressive puberty in siblings with MC4R p. Arg165Gln: longitudinal follow-up and gonadotropin-releasing hormone agonist treatment.

authors

- Huang H
- Yang Y
- Xiong X
- Zhang D
- Yang L
- Xie L

journal: Frontiers in endocrinology

journalAbbr: Front Endocrinol (Lausanne)

year: 2026

pubdate: 2026

volume: 17

issue:

pages: 1799027

abstract: BACKGROUND: Pathogenic variants in melanocortin-4 receptor (MC4R) are the most common cause of nonsyndromic monogenic obesity. However, links between MC4R -related obesity and pubertal tempo, and the long-term endocrine-metabolic trajectory in East Asian children, remain insufficiently characterized. Objective: To describe the longitudinal endocrine and metabolic features and treatment response of two siblings carrying a pathogenic MC4R variant who developed rapidly progressive puberty with central activation. CASE REPORT: We report two Chinese siblings with severe early-onset obesity and accelerated pubertal tempo. Trio-based whole-exome sequencing with segregation analysis identified a heterozygous MC4R variant (NM_005912.3: p.Arg165Gln). The proband,a girl, initially presented with premature thelarche, followed by rapid progression with advanced bone age and increased growth velocity. A GnRH stimulation test demonstrated a pubertal luteinizing hormone response (peak LH 6.61 mIU/mL). GnRH agonist (GnRHa) therapy stabilized pubertal progression until discontinuation, after which spontaneous puberty resumed and menarche occurred at 13 years 6 months. At 11 years 8 months, metabolic evaluation revealed impaired glucose tolerance with hyperinsulinemia/insulin resistance and hypertriglyceridemia.Short-term metformin therapy, together with intensified lifestyle intervention, was followed by sustained weight reduction and durable glycemic normalization. At the most recent follow-up (14 years 5 months), height was 162 cm (SDS + 0.49), weight was 55 kg (SDS + 0.77), BMI was 20.96 kg/m 2 (SDS + 0.66), and fasting blood glucose remained within the normal range. The younger brother showed concordant severe obesity with rapid pubertal progression, pubertal basal gonadotropins, and a strongly positive GnRH stimulation test (peak LH 50.82 mIU/mL; peak LH/FSH ratio 1.30). Although GnRHa was initiated relatively late, at 10 years 3 months, treatment was undertaken not only because of rapid pubertal progression, persistent bone-age advancement, and concern for compromised height potential, but also because of marked psychosocial distress and strong parental preference for active intervention. Biochemical suppression was achieved at 10 years 6 months (peak LH 0.88 mIU/mL), with stable clinical staging thereafter. CONCLUSION: Longitudinal follow-up of siblings carrying the pathogenic MC4R p. Arg165Gln variant highlights two clinical priorities in severe pediatric obesity, timely recognition and individualized management of rapidly progressive centrally activated puberty, and sustained guideline-informed surveillance and treatment of cardiometabolic comorbidities through long-term lifestyle intervention.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13175789/

Record 34

pmcid: PMC13171438

pmid: 42147082

doi: 10.1210/jcemcr/luag097

title: Central precocious puberty in partial androgen insensitivity syndrome: a 9-year follow-up.

authors

- Chen K
- Yao H

journal: JCEM case reports

journalAbbr: JCEM Case Rep

year: 2026

pubdate: 2026 Jun

volume: 4

issue: 6

pages: luag097

abstract: Androgen insensitivity syndrome (AIS) is a group of complex clinical symptoms caused by X-linked genetic defects. It results from inactivating mutations in the androgen receptor (AR) g ene, leading to hormone resistance in individuals with an XY karyotype despite normal androgen synthesis. We present a case of a 2-year-1-month-old boy who initially showed signs of incomplete masculinization of the external genitalia and underwent transverse preputial island flap urethroplasty during childhood. At the age of 7, he presented with gynecomastia, accompanied by accelerated growth and advanced bone age, with significantly elevated serum sex hormone levels. During this period, epididymitis recurred repeatedly. Genetic testing indicated a pathogenic mutation in the androgen receptor gene. The diagnosis of partial AIS with central precocious puberty was established, and treatment was initiated sequentially with oral letrozole and tamoxifen. The patient underwent multiple surgical procedures and experienced distress related to breast development. A review of the literature indicates that most patients with AIS present with delayed puberty; to date, only 2 cases of AIS combined with central precocious puberty have been reported. This case highlights diagnostic challenges, treatment considerations, and the importance of long-term multidisciplinary care, and further delineates the clinical phenotype.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13171438/

Record 35

pmcid: PMC13236702

pmid: 42254560

doi: 10.53045/jprs.2024-0084

title: Circumareolar Skin Reduction versus Long Scar in Surgical Treatment of High-Grade Gynecomastia.

authors

- El Shamy A
- Omar A
- El Sakka D
- El Nahas M

journal: Journal of plastic and reconstructive surgery

journalAbbr: J Plast Reconstr Surg

year: 2026

pubdate: 2026 May 27

volume: 5

issue: 3

pages: 158-168

abstract: OBJECTIVE: The objective of this study was to compare the surgical treatment of gynecomastia using circumareolar excision versus transverse skin excision. METHODS: Thirty-six male patients with unilateral or bilateral gynecomastia participated in this prospective randomized study at our institution. Participants were divided into two groups: Group A received circumareolar skin excision, and group B received transverse skin excision. Patients were evaluated for surgical and postoperative complications, patient satisfaction, and cosmetic outcomes. RESULTS: There were no intraoperative complications or blood transfusions. The average operative time was 150 minutes for group A and 110 minutes for group B. Hospital stays ranged from one to two days. Both groups had a 5.56% incidence of hemostasis-related issues and wound infections. Seromas appeared in 11.11% of cases and resolved spontaneously. Wound dehiscence was observed in 16.67% of group A and 5.56% of group B, with no statistically significant difference. Hypertrophic scars occurred in 16.67% of group B, while none were reported in group A (p = 0.07). CONCLUSIONS: The circumareolar technique requires longer surgical time and may lead to a less optimal chest contour. However, it provides superior cosmetic results for the nipple-areola complex. Both techniques demonstrate comparable rates of postoperative complications.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13236702/

Record 36

pmcid: PMC13599737

pmid: 42775998

doi: 10.4274/ejbh.galenos.2026.2026-1-12

title: Clinical Management After Rupture of Silicone Gel-Filled Breast Implants in a Surgically Treated Cohort: A Retrospective Study.

authors

- Tran MT
- Chotarova M
- Harsanyi S
- Mitevova D
- Bohac M

journal: European journal of breast health

journalAbbr: Eur J Breast Health

year: 2026

pubdate: 2026 Sep 23

volume: 22

issue: 4

pages: 434-440

abstract: OBJECTIVE: Breast implant rupture is clinically significant complication that is often asymptomatic making diagnosis challenging. The primary objective of this study was to describe the clinical presentation, preoperative imaging findings, and operative findings in this surgically selected cohort. A secondary objective was to assess the apparent concordance of magnetic resonance imaging (MRI) and ultrasound findings with intraoperative rupture status in this surgically selected cohort. Additional exploratory analyses examined symptom categories, implant age, and implant placement in relation to operative and imaging findings. MATERIALS AND METHODS: This retrospective observational study evaluated a cohort of women with suspected or confirmed silicone breast implant rupture who underwent operative management at the Department of Plastic Surgery, Faculty of Medicine, Comenius University, University Hospital Bratislava, Ružinov, between 2020 and 2025. Because only patients who proceeded to surgery were included, the cohort represents a selected referral population rather than an unselected surveillance population. RESULTS: The mean patient age was 50.6 years, with a mean implant age of 14.7 years. Within this surgically selected cohort, MRI demonstrated higher apparent sensitivity and positive predictive value than those of ultrasound; however,

diagnostic performance varied across subgroups and the findings should be interpreted cautiously because of the small sample size and selection bias. In patients with local symptoms, the apparent diagnostic performance was lower, particularly for ultrasound. CONCLUSION: Breast implant rupture showed heterogeneous clinical presentations in this retrospective cohort of surgically treated patients, ranging from asymptomatic, imaging-detected rupture to cases associated with local symptoms, capsular contracture, or systemic complaints. MRI showed a higher apparent concordance with intraoperative rupture status than ultrasound in this selected cohort; however, the findings are limited by selection and verification bias. Subgroup analyses were exploratory, and clinical management should remain individualized; prospective studies are needed to guide evidence-based imaging and treatment strategies.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13599737/

Record 37

pmcid: PMC13201066

pmid: 42175417

doi: 10.1097/MD.0000000000048669

title: Comparative adverse event reporting patterns of desogestrel versus norethisterone in third-generation combined oral contraceptives: A disproportionality analysis of the FAERS satabase.

authors

- Li J
- Qin L
- Fu Y
- Wu Q
- Fang X
- Liu H

journal: Medicine

journalAbbr: Medicine (Baltimore)

year: 2026

pubdate: 2026 May 22

volume: 105

issue: 21

pages: e48669

abstract: The aim was to investigate the adverse event reporting patterns of 2 commonly used combined contraceptives (desogestrel/ethinyl estradiol (DES) and norethisterone/ethinyl estradiol (NOR)) using the Food and Drug Administration Adverse Event Reporting System database. We selected data from the first quarter of 2004 to the second quarter of 2024 from the Food and Drug Administration Adverse Event Reporting System database for disproportionate analysis. We performed signal detection and comparative analysis using the reporting odds ratio, proportional reporting ratio, Bayesian Confidence Propagation Neural Network, and Empirical Bayes Geometric Mean for adverse events (AEs) related to DES, and NOR. From January 1, 2004 to June 30, 2024, a total of 35,83,091 reports related to drug AEs were extracted. Among these, 1177 reports were associated with DES, involving 4057 AEs; 1677 reports were associated with NOR, involving 4353 AEs. In the category of reproductive system and breast disorders, the reporting association for abnormal withdrawal bleeding was stronger for DES, while withdrawal bleeding and hypomenorrhoea were more frequently reported with NOR. In pregnancy, puerperium, and perinatal conditions, signals for complications of delivery and labor were more prominent in patients using DES. Conversely, NOR showed stronger associations with reports of contraceptive failure (pregnancy during oral contraceptive use and pregnancy while using contraceptives). In vascular disorders, vena cava thrombosis and splenic infarction were reported more frequently in patients using NOR, while arterial thrombosis and embolism were more commonly reported in those using DES. Subgroup analyses showed that women aged 18 to 40 years had a greater number of preferred terms and signal strength

in the aforementioned 3 major systems. In this analysis, DES showed stronger reporting signals for arterial thrombosis, perinatal complications, and abnormal withdrawal bleeding, while NOR was more associated with vena cava thrombosis, contraceptive failure, withdrawal bleeding, and breast enlargement. Clinicians may pay particular attention to women of reproductive age, specifically those aged 18 to 40 years.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13201066/

Record 38

pmcid: PMC12967101

pmid: 41798405

doi: 10.7759/cureus.103021

title: Comparison of Antipsychotics in the Treatment of COVID-19-Induced First-Episode Psychosis: A Review of Case Studies.

authors

- Singh G
- Hartnett R
- Silva BM
- Fekrat SMM
- Prasad S
- Gill G
- Gunturu S

journal: Cureus

journalAbbr: Cureus

year: 2026

pubdate: 2026 Feb

volume: 18

issue: 2

pages: e103021

abstract: This study aims to systematically review COVID-19-associated first-episode psychosis cases, comparing antipsychotic selection, dosing strategies, treatment response timelines, adverse effects, and relapse rates to inform evidence-based pharmacological management. We conducted a structured narrative review of published case reports and series describing COVID-19-Induced first-episode psychosis treated with antipsychotics. A comprehensive search of PubMed and Google Scholar (Jan 2020-Apr 2023) identified 42 eligible cases based on predefined inclusion/exclusion criteria. Data were extracted using a standardized template and summarized descriptively due to clinical heterogeneity. Variables included demographics, psychiatric features, antipsychotic(s) used, clinical course, and outcomes. First-episode psychosis (FEP) was higher in males (24, 57.1%) and the 30-39 age group (10, 23.8%). Olanzapine was the most commonly used single antipsychotic (6, 28.6%), while the combination of haloperidol and aripiprazole was the most frequently used antipsychotic regimen (4, 19.0%). Atypical antipsychotics were preferred (54.8%), with olanzapine (23, 54.8%) being the most commonly used at a mean dose of 10.9 mg/day. Reported side effects included fatigue, weight gain, akathisia, leukocytosis, and QT-interval prolongation (5, 11.9%), with a relapse rate of (2, 4.8%). This review evaluates the treatment methods for COVID-19 FEP and develops a deeper understanding of various antipsychotics used in managing psychosis and its outcomes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12967101/

Record 39

pmcid: PMC13471610

pmid: 42168959

doi: 10.1186/s12893-026-03823-1

title: Comprehensive use of combined surgical techniques performing liposuction, round-block periareolar de-epithelialization, and complete subcutaneous breast tissue resection approach in grade III gynecomastia: a review of 82 patients.

authors

- Soltani H
- Nikeghbalian Z
- Eskandari A
- Ahmadinejad M
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- Karimi S
- Amadeh N
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journal: BMC surgery

journalAbbr: BMC Surg

year: 2026

pubdate: 2026 May 21

volume: 26

issue: 1

pages:

abstract: BACKGROUND: Gynecomastia is associated with significant psychological and cosmetic concerns. OBJECTIVE: Surgical approaches for gynecomastia that are minimally invasive, associated with few complications, allow for rapid recovery, and result in high patient satisfaction are highly valuable. This study aims to evaluate the aesthetic efficacy and complications of accumulated techniques for treating patients with grade III gynecomastia. METHODS: The preferred strategy includes a combination of enhanced liposuction, round-block de-epithelialization, complete subcutaneous dissection of fibroglandular tissue, and reconstruction methods. Demographic characteristics, complications, satisfaction questionnaires with a 5-point Likert scale, and postoperative quality of life with WHO-QOL-BREF questionnaire were evaluated during follow-up. RESULTS: This study included 82 male patients with gynecomastia grade III (including 3 transgender patients) with a mean age of 30.18 ± 7.60 and the mean BMI of 30.97 ± 3.96. The overall postoperative satisfaction of the patients was 78%. Most patients reported being satisfied or very satisfied with the breast appearance (65.8%), symmetry (75.6%), skin tightness (73.2%), excessive skin resection (64.6%), healing of the incision (62.2%), and contour irregularity (75.6%). Seroma (12.2%), and edema (31.7%) were the most reported postoperative complications which gradually resolved, whereas other complications were insignificant such as bleeding (2.4%), subcutaneous hematoma (3.7%), NAC necrosis (3.7%), and infection (2.4%). No evidence of hemo/pneumothorax, intrathoracic organ damage, and fat embolism, were observed. Regarding scar characteristics, 1.2% had prominent scars and 86.6% had small or hidden scars. Regarding incision characteristics, wound disruption occurred in 8.5% of the cases, whereas 89% of the patients had minimal and good length incisions. CONCLUSION: Enhanced liposuction, round-block periareolar de-epithelialization, and complete subcutaneous breast tissue resection approach in patients with grade III gynecomastia and severe ptosis is a feasible, and safe approach with rapid recovery, hidden scars and acceptable patient satisfaction. TRIAL REGISTRATION: Not applicable. This study is a retrospective observational case series and does not require trial registration.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13471610/

Record 40

pmcid: PMC13547029

pmid: 42707248

doi: 10.3389/fendo.2026.1910860

title: Could refeeding-associated gynaecomastia occur during recovery from anorexia nervosa? A hypothesis-generating review.

authors

- Mousa AM
- Mohamed MMA
- Mahmoud-Katheer M

journal: Frontiers in endocrinology

journalAbbr: Front Endocrinol (Lausanne)

year: 2026

pubdate: 2026

volume: 17

issue:

pages: 1910860

abstract: International andrology guidance discusses refeeding after starvation as a clinical setting associated with gynaecomastia, based on reports from starvation-refeeding settings outside anorexia nervosa (AN). A targeted search of PubMed, PsycINFO, and Google Scholar, last conducted on 27 July 2026, identified no indexed reports in male patients recovering from AN. We therefore advance the testable hypothesis that transient glandular breast tissue may occasionally occur but remain unrecognised during nutritional recovery; the available evidence does not establish occurrence, prevalence, or clinical significance in AN. Five endocrine transitions provide biological plausibility: non-uniform recovery of the hypothalamic-pituitary-gonadal axis, expansion of the adipose tissue compartment in which aromatase is expressed, a transient leptin rise, possible preservation of adrenal androgen precursor availability, and recovery of insulin-like growth factor 1. Direct human data in male AN support parts of the endocrine trajectory, whereas the proposed link to breast tissue relies largely on indirect human evidence and preclinical studies. Potential under-recognition could arise from misclassification as pseudogynaecomastia, patient-side under-reporting, and follow-up that ends before later endocrine recovery; no clinical detection window has been established. We propose retrospective review, prospective surveillance extending beyond acute refeeding, targeted endocrine evaluation of confirmed glandular tissue, and multicentre studies of prevalence, risk factors, natural history, psychological effects, and treatment engagement.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13547029/

Record 41

pmcid: PMC13400497

pmid: 42502652

doi: 10.1007/s13139-025-00950-0

title: COVID-19 Vaccination Induced Gigantomastia and Breast Lymphedema Visualized on Breast Lymphoscintigraphy and Single-Photon Emission Computed Tomography/Computed Tomography.

authors

- Kim JY
- Hwang JW
- Park S
- Woo KJ
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journal: Nuclear medicine and molecular imaging

journalAbbr: Nucl Med Mol Imaging

year: 2026

pubdate: 2026 Aug

volume: 60

issue: 4

pages: 423-425

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13400497/

Record 42

pmcid: PMC13491697

pmid: 42289661

doi: 10.1186/s12884-026-09458-7

title: Current situation and influencing factors of knowledge, attitude and practice of breast engorgement among postpartum women: a cross-sectional study.

authors

- Li A
- Tu S
- Guo Q
- Gao J
- Liu Q

journal: BMC pregnancy and childbirth

journalAbbr: BMC Pregnancy Childbirth

year: 2026

pubdate: 2026 Jun 15

volume: 26

issue: 1

pages:

abstract: OBJECTIVE: To understand the current situation of knowledge, attitude and practice of breast engorgement in postpartum women and analyze its influencing factors. METHODS: From August 2019 to December 2019, by using the method of convenient sampling, 530 postpartum women in the department of obstetrics in 3 tertiary hospitals were investigated with the breast engorgement knowledge, attitude and practice and general data questionnaire. RESULTS: The scores of breast engorgement of knowledge, attitude, practice and total KAP were (8.90 ± 3.17), (33.83 ± 3.54), (31.67 ± 1.41) and (74.41 ± 12.26). The results of regression analysis showed that participation in school during pregnancy and knew breast engorgement were influencing factors of breast engorgement knowledge score. Graduate and above (maternity), participation in school during pregnancy and vaginal delivery were influencing factors of attitude score towards breast engorgement. Artificial feeding and mixed feeding were influencing factor of practice score of breast engorgement. Participation in school during pregnancy, knew breast engorgement, artificial feeding, mixed feeding and commercial and service personnel (spouse) were influencing factors of total score of KAP of breast engorgement ($P<0.05$). CONCLUSION: The level of knowledge about breast engorgement is low, the attitude towards breast engorgement is positive, and the practice to breast engorgement need to be improved. Health care workers should formulate and implement targeted nursing education and intervention to improve the level of knowledge, attitude and practice of breast engorgement in order to reduce the incidence of breast engorgement.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13491697/

Record 43

pmcid: PMC13613354

pmid: 41104791

doi: 10.1111/jdv.70115

title: Deep and disseminated dermatophytosis in immunocompromised populations-A systematic review.

authors

- Gupta AK
- Wang T
- Susmita
- Saunte DML
- Hay R
- Piguet V

journal: Journal of the European Academy of Dermatology and Venereology : JEADV

journalAbbr: J Eur Acad Dermatol Venereol

year: 2026

pubdate: 2026 Oct

volume: 40

issue: 10

pages: 1648-1661

abstract: BACKGROUND: Dermatophytes-a group of obligate keratinophilic fungi that infect the skin, hair and nails-represent the most common cause of fungal infections worldwide. In immunocompromised individuals, a growing body of literature has documented rare instances of dermatophyte infections of the dermis and subcutaneous tissue, which can lead to deep dermatophytosis with systemic dissemination and pseudomycetoma as secondary complications. OBJECTIVES: To assess patient characteristics, risk factors, diagnostic evidence and treatment options for deep dermatophytosis. METHODS: A systematic review was conducted in June 2025 to identify patients presenting with deep dermatophytosis of the glabrous skin; all cases were validated by confirmatory testing per histopathologic examination and fungal culture. Majocchi's granuloma diagnoses were excluded. RESULTS: The search yielded 96 studies (1954-2025) documenting 134 patients. The number of reports has notably increased since the 21st century; however, diagnosis remains challenging due to possible dermatophyte adaptation in the dermal/subcutaneous environment. Trichophyton rubrum is the most frequently isolated pathogen. Acquired immunodeficiencies (e.g. renal transplant recipients) and primary immunodeficiencies (e.g. CARD9, STAT3 mutations) elevate the risk of deep dermatophytosis. Systemic dissemination, often accompanied by lymphadenopathy, was reported in 24 patients, with the most common being infection of the axillary and inguinal lymph nodes. Infections of the bones, lungs, and brain were also reported. Terbinafine (125-1000 mg/day) and itraconazole (100-400 mg/day), with adjunctive topical antifungals and surgical interventions, have been tried for deep dermatophytosis. The use of newer triazoles (voriconazole, posaconazole) should follow the principles of antifungal stewardship and therapeutic drug monitoring recommendations. Multidrug resistance is a potential concern in immunocompromised patients who will often require extended or lifelong treatment. CONCLUSIONS: Healthcare providers should be cognizant that dermatophytosis may become invasive and disseminated in immunocompromised populations. Testing and early treatment should not be overlooked in these patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13613354/

Record 44

pmcid: PMC13031283

pmid: 41748694

doi: 10.1038/s41598-026-41646-5

title: Difference in breast size between the first and third trimesters of pregnancy and changes in a woman's body size, subcutaneous fat tissue, and biochemical parameters.

authors

- Tutkuviene J
- Gervickaite S
- Drasutis J
- Sakalauskaite I
- Simkunaite-Rizgeliene R
- Jablonskiene V
- Barkus A
- Jakimaviciene EM
- Karciauskaite D
- Kaminskas A
- Tutkus V
- Tutkus J
- Nedzinskiene L
- Cepuliene R
- Abaravicius JA
- Ramasauskaite D

journal: Scientific reports

journalAbbr: Sci Rep

year: 2026

pubdate: 2026 Feb 26

volume: 16

issue: 1

pages:

abstract: The greatest changes in the breasts occur during pregnancy, but during this time the entire woman's body also undergoes specific modification, however, only a few articles discuss breast changes during pregnancy. We aimed to analyse the variation of breast volume in relation to body size, subcutaneous fat tissue and biochemical parameters during pregnancy. The same female (N = 83) was studied twice - in the first and third trimester of pregnancy. Breast volume was calculated according to the formula of Kramer&Drexler (1981). Breast volumes varied greatly both at the ca. 10th week (544-3500 cm^3) and at ca. 35th week (689-3730 cm^3) of pregnancy. Only 35-67% (depending on the trimester) of women's breast volume categories (small, medium, large) matched their body fat categories (lean, average, obese). Breast size was mainly related to skinfolds located on the trunk and upper limbs. The largest changes in body parameters occurred in the group of small-breasted and thin women. In addition, the greatest increase in high-density cholesterol during pregnancy was seen in women with medium to large breasts (which may represent a positive effect of pregnancy), but higher levels of low-density lipoprotein cholesterol in early pregnancy were associated with a higher likelihood of larger breasts.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13031283/

Record 45

pmcid: PMC13175282

pmid: 42147594

doi: 10.7759/cureus.107037

title: Diffuse Large B-cell Lymphoma of the Breast Presenting 40 Years After Breast-Conserving Therapy: A Case Report.

authors

- Xanthopoulou G
- Barkolias C
- Theodorolea K
- Spyrou I
- Tasis NP

journal: Cureus

journalAbbr: Cureus

year: 2026

pubdate: 2026 Apr

volume: 18

issue: 4

pages: e107037

abstract: Breast-conserving surgery combined with radiotherapy has been established as a standard therapeutic option for the management of early-stage breast cancer. Although radiation-associated tumors most frequently include sarcomas, lung cancer, and contralateral breast carcinoma, lymphoid malignancies arising within previously irradiated breast tissue are rarely reported. Breast lymphoma is a rare entity, and its occurrence following prior treatment for breast cancer may pose a diagnostic challenge as it can mimic recurrence of the primary malignancy. We report a case of diffuse large B-cell lymphoma of the breast in an 88-year-old woman presenting approximately 40 years after breast-conserving surgery and radiotherapy for invasive ductal carcinoma. The lesion was initially suspected to represent recurrent breast cancer based on clinical and imaging findings; however, histopathological and immunohistochemical evaluation confirmed lymphoma. Although the tumor arose within a previously irradiated field, a causal relationship with prior radiotherapy could be established and should be interpreted with caution. This case highlights the importance of considering alternative diagnoses in patients presenting with new breast lesions long after initial cancer treatment, particularly in elderly individuals.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13175282/

Record 46

pmcid: PMC13344099

pmid: 42422338

doi: 10.1093/asjof/ojag117

title: Do Textured Breast Implants Mitigate Malposition? A Systematic Review and Meta-Analysis.

authors

- Vonu PM
- Ahmed SH
- Gerhold C
- Usmani SUR
- Mast BA

journal: Aesthetic surgery journal. Open forum

journalAbbr: Aesthet Surg J Open Forum

year: 2026

pubdate: 2026

volume: 8

issue:

pages: ojag117

abstract: The widespread utilization of silicone implants for cosmetic augmentation and breast reconstruction has encouraged many innovative design characteristics since inception in 1962. Despite a multitude of studies, there exists limited collective evidence supporting textured implants' efficacy in reducing implant malposition. In this study, the authors present a systematic review and meta-analysis to assess all available data regarding textured devices associated with outcomes of implant malposition. A literature search was performed covering PubMed/Medline, Scopus, and Web of Science databases. Cohort studies assessing textured vs nontextured implants were included in the final review. Primary outcome data included rates of implant malposition, and secondary outcomes included capsular contracture, rippling, and infection. Nine studies met inclusion criteria, including 2011 patients undergoing breast reconstruction and 1229 undergoing cosmetic augmentation. Malposition occurred in 5.2% of breasts in the textured cohort and 8.9% in the smooth cohort; however, pooled analysis demonstrated no statistically significant difference between groups. No significant differences were observed for capsular contracture, seroma, or rippling. A higher rate of infection was observed in the textured cohort, although this finding was not consistent across all analyses. No cases of breast implant-associated anaplastic large cell lymphoma were reported. Review of existing literature failed to identify statistical superiority in textured implants for reducing malposition. Interpretation is limited by retrospective design and heterogeneity among included studies. Level of Evidence: 3 (Therapeutic) For image description, please refer to the figure legend and surrounding text.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13344099/

Record 47

pmcid: PMC13118824

pmid: 42075061

doi: 10.3390/nu18081248

title: Does Functionality Appreciation Mediate the Relationship Between Breastfeeding Attitudes and Breastfeeding Intentions?

authors

- Di Gesto C
- Spinoni M
- Grano C

journal: Nutrients

journalAbbr: Nutrients

year: 2026

pubdate: 2026 Apr 15

volume: 18

issue: 8

pages:

abstract: BACKGROUND: This study aimed to contribute to the growing empirical interest in the role of positive body image in the context of breastfeeding. RESEARCH AIM: We examined the association between positive attitudes toward breastfeeding and breastfeeding intentions among postpartum women and investigated the mediation of functionality appreciation. METHOD: A total of 305 women who had given birth within the past 1 to

3 months (M = 34.11 years) participated in the study. Women completed a questionnaire assessing breastfeeding attitudes, appreciation of breastfeeding functionality, breastfeeding intentions, previous breastfeeding experience, as well as Body Mass Index and sociodemographic and obstetric characteristics. A mediation model was used to examine direct and indirect associations between attitudes toward breastfeeding, breastfeeding intentions, and functionality appreciation. RESULTS: Results showed significant associations between positive attitudes towards breastfeeding, breastfeeding intentions, and functionality appreciation. Positive attitudes toward breastfeeding were positively associated with breastfeeding intentions ($\beta = 0.63$, $p < 0.001$) and with functionality appreciation ($\beta = 0.51$, $p < 0.001$), with functionality appreciation accounting for a significant indirect association ($\beta = 0.21$, 95% CI [0.14, 0.29]). Finally, previous breastfeeding experience was positively associated with breastfeeding intentions ($\beta = 0.15$, $p < 0.001$). CONCLUSIONS: This study marks the initial attempt to examine the significance of functionality appreciation in postpartum women, highlighting potential associations between breastfeeding attitudes and breastfeeding intentions. These findings may offer preliminary insights for future research and for informing the development of targeted interventions, although further evidence from more diverse populations is needed.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13118824/

Record 48

pmcid: PMC13424140

pmid: 42533012

doi: 10.1038/s41598-026-63711-9

title: Effect of pilates exercises plus breastfeeding positioning adjustment on artificial intelligence-assessed craniovertebral angle in breastfeeding women with forward head posture: a randomized controlled trial.

authors

- El Blasy DO
- El-Din Mahmoud LS
- Abdulaziz KS
- El-Taweel AA
- Osman DA

journal: Scientific reports

journalAbbr: Sci Rep

year: 2026

pubdate: 2026 Jul 30

volume: 16

issue: 1

pages:

abstract: Forward head posture (FHP), considered a neuromusculoskeletal disorder, includes those associated with breastfeeding. Pilates is recognized for improving posture, flexibility, and core strength. To investigate the effect of Pilates on craniovertebral angle (CVA), cervical spine mobility, pain, neck disability, fatigue, breastfeeding self-efficacy, quality of life (QOL), and infant anthropometric measurements in breastfeeding women with FHP. Seventy-four breastfeeding women with FHP were randomized into two equal groups. The control group (n = 37) received breastfeeding positioning adjustment only, while the intervention group (n = 37) received breastfeeding positioning adjustment in addition to Pilates exercises. The Primary outcomes included CVA assessed using Artificial Intelligence Posture Evaluation and Correction System (APECS) and cervical range of motion (ROM), while secondary outcomes included Visual Analogue Scale for pain (VAS-Pain), Neck Disability Index (NDI), Fatigue Severity Scale (FSS), Breastfeeding Self-Efficacy Scale-Short Form (BSES-SF), QOL assessed using Short Form-36 Health Survey (SF-36), and infant anthropometric measurements (weight and length). All outcomes were assessed at the Outpatient Clinic of the Physical Therapy Department, Suez Canal Authority Hospital, Port Said, Egypt, at baseline and after the 8-week intervention. Mixed-design

MANOVA demonstrated significant improvements from pre- to post-intervention in both groups across all measured outcomes (all $p < 0.001$). However, the study group exhibited significantly greater improvements than the control group in all outcome measures (all $p < 0.001$). The magnitude of these effects was large to very large, with partial eta-squared values ranging from 0.621 to 0.990. In breastfeeding women with FHP, Pilates exercises with breastfeeding positioning adjustment significantly improve CVA, cervical spine mobility, breastfeeding self-efficacy, QOL, and infant growth parameters, while reducing pain, neck disability, and fatigue.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13424140/

Record 49

pmcid: PMC13358330

pmid: 42445471

doi: 10.3389/fsurg.2026.1822281

title: Effectiveness and safety of periareolar micro-excision combined with liposuction for gynecomastia: a retrospective cohort of 177 patients.

authors

- Guo X
- Song B

journal: Frontiers in surgery

journalAbbr: Front Surg

year: 2026

pubdate: 2026

volume: 13

issue:

pages: 1822281

abstract: OBJECTIVE: Gynecomastia is common and carries substantial psychosocial burden; liposuction alone has limited efficacy for glandular removal. This study evaluated the effectiveness and safety of liposuction combined with periareolar micro-incision gland excision for the treatment of gynecomastia. METHODS: From January 2019 to April 2025, we treated 177 patients with gynecomastia. All underwent liposuction plus periareolar micro-incision gland excision. We performed a retrospective review of clinical records to assess long-term surgical outcomes, complications, and patient satisfaction. RESULTS: The median excised gland weight was 41.9 g (range, 8-180 g); the median lipoaspirate volume was 882 mL (range, 100-2,500 mL). The median operative time was 89.7 min (range, 60-120 min). Complications included seroma in 15 patients, hematoma in 3 patients within 6 h postoperatively, infection in 3 patients, and transient cutaneous sensory changes in 4 patients. The mean patient-reported satisfaction score was 8.8/10. CONCLUSIONS: Liposuction combined with periareolar micro-incision gland excision is a technically feasible and cosmetically favorable treatment for gynecomastia. Although postoperative wrinkling or folding of the nipple-areolar complex may occur in patients with more severe deformities, the technique can still provide satisfactory aesthetic outcomes while keeping visible scarring to a minimum. Attention to uniform tumescent infiltration, meticulous hemostasis, dead-space control, and staged compression may further reduce early hematoma and seroma and help prevent postoperative asymmetry, particularly in patients with higher BMI.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13358330/

Record 50

pmcid: PMC13108662

pmid: 42040530

doi: 10.1097/GOX.0000000000007650

title: Eight Women With Autologous Fat Transfer and Deep Inferior Epigastric Perforator Flap Breast

Reconstruction: Which Do They Prefer, and Why?

authors

- van der Venne WBW
- Choudhury I
- de Wit T
- Heuts EM
- van der Hulst RRWJ
- Piatkowski AA

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2026

pubdate: 2026 Apr

volume: 14

issue: 4

pages: e7650

abstract: BACKGROUND: Autologous fat transfer (AFT) is emerging as an alternative autologous reconstruction postmastectomy. This study compared patient-reported outcomes of AFT and deep inferior epigastric perforator (DIEP) flap in women who underwent both procedures. METHODS: In this mixed-methods, cross-sectional study, 8 women meeting inclusion criteria completed BREAST-Q and BODY-Q surveys, of whom 6 subsequently participated in semistructured interviews. Quantitative data compared satisfaction, physical well-being, and sensation. Qualitative thematic analysis explored patient experiences. Additional clinical data were collected retrospectively. RESULTS: DIEP flap reconstruction showed superior scores across domains, with a statistically significant advantage in physical well-being (P = 0.029, Cohen d = 0.80). Satisfaction and sensation differences were small to moderate, with qualitative data revealing more nuanced experiences. Participants described DIEP flap reconstruction as a 1-time, more invasive procedure with longer recovery and visible scarring, whereas AFT was perceived as gradual, less traumatic, and associated with minimal scarring, albeit requiring multiple sessions and showing variable volume retention. Sensory recovery was limited in both. Decision-making was influenced by treatment availability and patient values, in which social support played a critical role. CONCLUSIONS: In this exploratory study of women who experienced both DIEP flap and AFT breast reconstruction, AFT appeared to yield satisfaction similar to DIEP flap reconstruction, but further research is required to validate these observations.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13108662/

Record 51

pmcid: PMC13590322

pmid: 42769172

doi: 10.3389/fphys.2026.1832015

title: Electrical stimulation and biofeedback combined with pelvic floor muscle training for pelvic floor dysfunction in postpartum women: a systematic review and meta-analysis.

authors

- Huang X
- Hu Y
- Hu F
- Li C
- Huang X

- Ai Y

journal: Frontiers in physiology

journalAbbr: Front Physiol

year: 2026

pubdate: 2026

volume: 17

issue:

pages: 1832015

abstract: BACKGROUND: Postpartum pelvic floor dysfunction (PFD) manifests as decreased pelvic floor muscle strength (PFMS), stress urinary incontinence (SUI), pelvic organ prolapse (POP), and impaired sexual function. Pelvic floor muscle training (PFMT) is the first-line therapy but often limited by poor muscle perception and adherence. Electrical stimulation (ES) and biofeedback (BF) may enhance outcomes, yet their combined effect with PFMT has not been systematically quantified. METHODS: Following PRISMA 2020 guidelines, PubMed, Embase, Web of Science, Cochrane Library, CNKI, WanFang, VIP, and SinoMed were searched to July 9, 2026. Randomized controlled trials (RCTs) comparing ES+BF+PFMT with PFMT alone were included. The population was restricted to women within 12 months postpartum. Outcomes were PFMS, SUI incidence, POP severity, and sexual function. Pooled mean differences (MD) or risk ratios (RR) with 95% confidence intervals (CI) were calculated. Subgroup and sensitivity analyses were performed, publication bias was assessed for outcomes with at least 10 studies, and the certainty of evidence was evaluated using GRADE. RESULTS: Twenty-nine RCTs were included. ES+BF+PFMT significantly improved PFMS (MD = 0.85, 95% CI: 0.69-1.01), reduced SUI incidence (RR = 0.36, 95% CI: 0.26-0.49), lowered POP-Q stage (MD=-0.29, 95% CI: -0.39 to -0.20), and enhanced sexual function (MD = 4.38, 95% CI: 3.41-5.34). Exploratory subgroup analyses suggested that Kegel training duration may contribute to PFMS heterogeneity. GRADE assessment indicated moderate certainty for SUI incidence, low certainty for PFMS and sexual function, and very low certainty for POP severity. CONCLUSIONS: Compared with PFMT alone, ES+BF+PFMT may provide additional benefits for postpartum PFD rehabilitation within the first year, with the most consistent evidence observed for reducing SUI incidence. Potential benefits were also observed for PFMS and sexual function, whereas the POP-Q findings should be interpreted with particular caution because the certainty of evidence was very low. The generalizability of these findings is limited because all included studies were conducted in China. SYSTEMATIC REVIEW REGISTRATION: https://www.crd.york.ac.uk/PROSPERO/view/CRD420251134152, identifier CRD420251134152.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13590322/

Record 52

pmcid: PMC13382972

pmid: 42421559

doi: 10.1080/14796694.2026.2677774

title: Enzalutamide in biochemically recurrent prostate cancer: clinical implications from EMBARK for real-world practice.

authors

- Freedland SJ
- Shore ND

journal: Future oncology (London, England)

journalAbbr: Future Oncol

year: 2026

pubdate: 2026 Jun

volume: 22

issue: 14

pages: 1601-1612

abstract: The fourth episode in this podcast series features a discussion between the two principal investigators from the phase III EMBARK trial (NCT02319837). EMBARK showed that in patients with prostate cancer with high-risk biochemical recurrence, enzalutamide plus leuprolide (enzalutamide combination) and enzalutamide monotherapy significantly improved metastasis-free survival compared with leuprolide alone. Furthermore, overall survival was significantly longer with enzalutamide combination compared with leuprolide alone. No new safety signals were observed, and health-related quality of life was maintained with enzalutamide combination and monotherapy (as determined by patient-reported outcomes). In this episode, the speakers discuss some of the results from EMBARK in more detail and provide their expert opinion on what the findings mean for clinical practice in the real world. They also answer some of the frequently asked questions that they receive about the study from people around the world, including discussion of whether the results apply regardless of primary therapy, if intermittent therapy is a possibility, and how the findings from EMBARK should be applied with respect to the availability of different imaging technologies.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13382972/

Record 53

pmcid: PMC13238062

pmid: 42021296

doi: 10.1186/s12917-026-05499-4

title: Ethnozoological study of medicinal animals used in traditional healthcare in Andracha District, Ethiopia.

authors

- Sirna AM
- Gadisa T
- Awoke A

journal: BMC veterinary research

journalAbbr: BMC Vet Res

year: 2026

pubdate: 2026 Apr 22

volume: 22

issue: 1

pages:

abstract: BACKGROUND: Medicinal animals are integral to traditional healthcare worldwide, yet ethnozoological knowledge in many Ethiopian regions remains under-documented. This study documented the diversity of medicinal animals, associated cultural knowledge, preparation methods, and conservation implications among communities in Andracha District, Ethiopia. METHODS: A cross-sectional ethnobiological survey was conducted from January to August 2025 using semi-structured interviews, focus group discussions (FGDs), and field observations. Sixty-five informants participated, including 25 key informants purposively selected and 40 general participants randomly chosen. FGDs refined data collection instruments and validated traditional knowledge. Quantitative indices Relative Frequency of Citation (RFC), Informant Consensus Factor (ICF), Fidelity Level (FL), and Zoological Ethnoknowledge Index (ZEI) summarized knowledge patterns, species importance, and cultural reliability. Jaccard Similarity Index (JSI) and Rahman's Similarity Index (RSI) assessed cross-community similarity. Statistical analyses, including t-tests, ANOVA, and linear regression, explored variations across gender, age, education, and experience using R software. RESULTS: A total of 52 medicinal animal species were documented in Andracha District. Bos taurus, Apis mellifera, and Halictus scabiosae exhibited the highest RFC values. ICF ranged from 0.60 to 0.97, with the strongest agreement observed for respiratory and gastrointestinal ailments. The highest FL was recorded for Capra aegagrus hircus (FL = 83.3%), followed by Apis mellifera (FL = 77.8%). Knowledge of medicinal animals varied significantly with age, gender, literacy, and

healer experience (P < 0.05). Major threats to medicinal fauna included agricultural expansion, overgrazing, and overexploitation, while indigenous conservation practices such as sacred forest protection and community stewardship were actively maintained. CONCLUSION: Medicinal animals are crucial for healthcare, cultural identity, and livelihoods in Andracha District. Environmental pressures and generational gaps threaten both species and associated knowledge. Strengthening community-based conservation, sustainable harvesting, and intergenerational knowledge transmission is essential to safeguard this biocultural heritage. The study documents traditional practices but does not endorse ingestion or application of potentially hazardous animal products.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13238062/

Record 54

pmcid: PMC13285365

pmid: 42312740

doi: 10.2340/1651-226X.2026.45677

title: Evaluation of 3D-CRT vs. VMAT for radiotherapy of whole breast with simultaneous integrated boost.

authors

- Johansson G
- Kojoj R
- Surdashi S
- Olin M
- Fredén E
- Sen P
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- Wendt C

journal: Acta oncologica (Stockholm, Sweden)

journalAbbr: Acta Oncol

year: 2026

pubdate: 2026 Jun 18

volume: 65

issue:

pages: 543-552

abstract: BACKGROUND AND PURPOSE: This study evaluates volumetric modulated arc therapy (VMAT) as a potential alternative to three-dimensional conformal radiotherapy (3D-CRT) for whole-breast radiotherapy (RT) with simultaneous-integrated boost (SIB). Patient/material and methods: Ten left-sided breast cancer patients previously treated in our institution (whole breast with SIB) were selected. Clinical plans were generated using tangential field-in-field 3D-CRT technique. VMAT plans were retrospectively created. Dosimetric evaluation was performed according to our clinical guidelines. A robustness evaluation of the 3D-CRT and VMAT plans against simulated anatomical and setup uncertainties was also performed. Differences between techniques were analysed using a two-sided Wilcoxon signed-rank test with a significance level of 0.05. RESULTS: Both the 3D-CRT and VMAT plans fulfilled the clinical requirements for target volumes and organs at risk. VMAT plans showed a median increase in heart mean dose of 10.8%, compared to 3D-CRT (p < 0.05). Heart maximum dose was reduced by up to 56.7% with VMAT (p < 0.05), offering a clinically meaningful advantage. For the ipsilateral lung, no significant difference in mean dose was observed, V16Gy was significantly lower and V4Gy was significantly increased with VMAT. VMAT plans improved dose conformity to the boost planning target volume, (p < 0.05). Median monitor units (MU) values of 371 MU (359-466 MU) vs 927 MU (752-1018 MU) were obtained with 3D-CRT and VMAT. VMAT remained robust to simulated uncertainties for whole-breast but showed greater sensitivity in the boost volume. INTERPRETATION: VMAT was a feasible alternative to 3D-CRT

for whole-breast RT with SIB, improving boost dose-conformity and reducing maximum heart dose. However, VMAT requires careful dosimetric evaluation and robustness assessment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13285365/

Record 55

pmcid: PMC13052953

pmid: 41931341

doi: 10.1097/MD.0000000000048207

title: Evaluation of adverse event profiles for leuprolide and goserelin: Insights from the FDA adverse event reporting system following STROBE guidelines.

authors

- Zhou F
- Yang M
- Ji W
- Ma J
- Ren N
- Ren X

journal: Medicine

journalAbbr: Medicine (Baltimore)

year: 2026

pubdate: 2026 Apr 3

volume: 105

issue: 14

pages: e48207

abstract: Leuprolide and Goserelin, both gonadotropin-releasing hormone (GnRH) agonists, are commonly used for ovarian function suppression and the management of hormone-dependent cancers. However, systematic comparisons of their adverse event (AE) profiles based on real-world pharmacovigilance data remains limited. This study aimed to compare the safety signals associated with Leuprolide and Goserelin using the the U.S. Food and Drug Administration AE Reporting System (FAERS), thereby providing insights into their distinct AE patterns. Food and Drug Administration Adverse Event Reporting System reports from the first quarter of 2004 to the third quarter of 2024 were extracted and standardized. Disproportionality signal detection was performed using the reporting odds ratio, proportional reporting ratio, chi-square ($\chi 2$) test, and Bayesian confidence propagation neural network. Significant signals were defined as reporting odds ratio lower 95% CI >1, proportional reporting ratio ≥2 with $\chi 2$ ≥4, or Bayesian confidence propagation neural network information component (IC)-2SD >0. Key signals were compared between Leuprolide- and Goserelin-related reports. A total of 12,418,989 AE reports were identified, including 64,324 Leuprolide-related and 4253 Goserelin-related cases. Both drugs showed strong signals for "prostatic specific antigen increased" (Leuprolide: $\chi 2$ = 84,009.65, IC-2SD = 5.42; Goserelin: $\chi 2$ = 820.31, IC-2SD = 3.38), "blood testosterone increased" (Leuprolide: $\chi 2$ = 22,845.52, IC-2SD = 5.56; Goserelin: $\chi 2$ = 290.23, IC-2SD = 2.28), and "prostatic specific antigen abnormal" (Leuprolide: $\chi 2$ = 51,615.98, IC-2SD = 6.46; Goserelin: $\chi 2$ = 422.43, IC-2SD = 2.16). Additional strong signals, such as "prostate cancer metastatic" and "hot flush," were consistently detected in both groups. Notably, the distribution of AEs differed between drugs, suggesting drug-specific safety patterns. Leuprolide demonstrated strong associations with AEs in "Reproductive system and breast disorders," "Neoplastic benign and malignant conditions," and procedure-related categories, whereas Goserelin was more strongly linked to "Endocrine disorders." These findings highlight distinct pharmacovigilance profiles between the 2 drugs and provide clinically relevant evidence to support individualized risk monitoring in hormone-dependent cancer therapies.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13052953/

Record 56

pmcid: PMC12704238

pmid: 41063384

doi: 10.1002/pros.70074

title: Factors Associated With Male Breast Cancer Incidence Among Prostate Cancer Survivors: Real World Evidence From Veterans Affairs National Prostate Cancer Data Core.

authors

- Whyne EZ
- Choi SH
- Unni N
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- Dowell JE
- Jeon-Slaughter H

journal: The Prostate

journalAbbr: Prostate

year: 2026

pubdate: 2026 Feb

volume: 86

issue: 2

pages: 227-235

abstract: BACKGROUND: While male breast cancer incidence is rare, veteran status is found to be associated with increased risk, for incidence, a higher prevalence of male breast cancer patients was observed among male veteran prostate cancer survivors. This study leveraged the existing large-scale Veterans Affairs (VA) Prostate Cancer Data Core and examined factors associated with increased risk of male breast cancer incidence in veterans with prior prostate cancer diagnoses. METHODS: A retrospective cohort study of 1.3 million male veterans treated for prostate cancer at VA hospitals was conducted using the VA Prostate Cancer Data Core. Of these, 11,327 (0.86%) were newly diagnosed with male breast cancer on average 5.4 years post prostate cancer diagnosis. RESULTS: Multivariate Cox and competing risk model results found that younger onset age of prostate cancer (hazard ratio [HR] 0.97, 95% confidence interval [CI] 0.97-0.98), metastasized prostate cancer (HR 2.03, 95% CI 1.90-2.17), being Non-Hispanic (N-H) Black (HR 1.10, 95% CI: 1.05-1.15), radiation (HR 1.06, 95% CI: 1.02-1.11) and androgen deprivation therapy (ADT; HR 1.24, 95% CI 1.17-1.32) were associated with significantly increased risk of male breast cancer diagnosis. Prolonged use of cardiovascular disease (CVD) medications, furosemide (HR 1.51, 95% CI 1.39-1.63), spironolactone (HR 1.36; 95% CI 1.15-1.61), and digoxin (HR 1.50, 95% CI: 1.29-1.72), significantly increased risk for male breast cancer incidence. CONCLUSIONS: Younger age onset of prostate cancer, metastasized prostate cancer, prolonged use of CVD medications, radiation, and ADT cancer treatment were factors significantly associated with increased risk of being diagnosed with male breast cancer among male veteran prostate cancer survivors. The study findings may shed insights in cardio-oncology specific risk factors for male breast cancer among prostate cancer survivors.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12704238/

Record 57

pmcid: PMC12947447

pmid: 41620697

doi: 10.1186/s12884-026-08719-9

title: Factors influencing the adjustment of pregnant nurses in continuing to provide nursing care: a qualitative

content analysis.

authors

- Shayestefar M
- Asgari MR
- Jahanfar S
- Babamohamadi H

journal: BMC pregnancy and childbirth

journalAbbr: BMC Pregnancy Childbirth

year: 2026

pubdate: 2026 Jan 31

volume: 26

issue: 1

pages:

abstract: BACKGROUND: Nursing is a high-stress profession frequently marked by demanding workplace conditions. Pregnant nurses, in particular, face heightened pressure to balance work and family responsibilities while maintaining their professional obligations throughout pregnancy. Adjusting to the physical, emotional, and occupational challenges of pregnancy is a complex process with important implications for both maternal well-being and fetal development. This study aims to explore the factors that influence how pregnant nurses adjust to continuing to deliver nursing care during this critical period. METHODS: This study utilized the qualitative content analysis method proposed by Graneheim and Lundman. Eighteen participants were selected between May 2023 and August 2024 from healthcare centers affiliated with Semnan University of Medical Sciences, Iran, including fourteen pregnant nurses and four nursing managers (two head nurses and two supervisors). Based on the study's central research question, data collection tools included in-depth semi-structured interviews, field observations, and field notes. To ensure rich and comprehensive data, the study aimed to achieve maximum variation in age, educational level, work departments, and different stages of pregnancy. Data management was conducted using MAXQDA software, version 2020. RESULTS: Data analysis yielded three themes, including "Health Concerns," "Thoughts and Past Experiences," and "Workplace Tensions in the Nursing Profession," comprising seven categories and twenty-two subcategories. CONCLUSION: Pregnant nurses' adjustment is influenced not only by pregnancy-related factors such as workplace conditions, fetal health, and the physical and mental well-being of the mother, but also by past personal experiences. Identifying and understanding the factors affecting pregnant nurses' adjustment and designing a mandatory training for all managers on Iranian pregnancy labor laws enables nursing managers to reduce occupational stressors. These programs could be directed toward creating a system of non-reciprocal behaviors, voluntary shift swaps for pregnant staff, and the reassignment of high-risk nurses with physical and psychological problems. Such initiatives may improve nurses' adjustment during pregnancy, the quality of patient care, and the health of both mother and fetus.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12947447/

Record 58

pmcid: PMC13005103

pmid: 41863367

doi: 10.1093/schbul/sbaf221

title: Female Sexual Dysfunction Induced by Second-Generation Antipsychotic Drugs-A Disproportionality Analysis Based on the FAERS Database.

authors

- Wang D
- Cao Y

- Wu L
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journal: Schizophrenia bulletin

journalAbbr: Schizophr Bull

year: 2026

pubdate: 2026 Mar 7

volume: 52

issue: 2

pages:

abstract: BACKGROUND: Second-generation antipsychotics (SGAs) are widely prescribed for psychiatric disorders but frequently cause sexual dysfunction, particularly in women-a side effect often underrecognized. This study assessed the association between SGAs and female sexual dysfunction using the US Food and Drug Administration Adverse Event Reporting System (FAERS). STUDY METHODS: We analyzed FAERS data from Q1 2004 to Q4 2023, identifying female sexual dysfunction cases using predefined MedDRA Preferred Terms. Nine SGAs were included: aripiprazole, quetiapine, olanzapine, risperidone, paliperidone, lurasidone, brexpiprazole, asenapine, and ziprasidone. Disproportionality analyses were conducted using frequentist (ROR, PRR) and Bayesian (BCPNN, MGPS) methods. Results were stratified by symptom type and prolactin-related drug effects. STUDY RESULTS: A total of 11 786 reports were identified, peaking in 2017. Women aged 19-41 years accounted for the largest subgroup (n = 4787, 40.6%). Antipsychotics accounted for 9 of the top 50 drugs linked to sexual dysfunction, with aripiprazole, quetiapine, and olanzapine most frequently reported. Aripiprazole was strongly associated with compulsive sexual behavior (ROR: 296.23) and hypersexuality. Risperidone and paliperidone were linked to decreased libido and anorgasmia. Prolactin-elevating drugs were associated with older age, intramuscular use, and more serious outcomes. CONCLUSION: This pharmacovigilance study highlights significant associations between specific SGAs and female sexual dysfunction. Disproportionality signals vary by drug, symptom type, and prolactin-related mechanisms. Clinicians should consider sexual side effects in treatment decisions and monitor patients accordingly. Further prospective studies are warranted.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13005103/

Record 59

pmcid: PMC13437946

pmid: 42552627

doi: 10.1002/nop2.70684

title: Frailty and Social Isolation of Elderly Prostate Cancer Patients: A Chain Mediating and Network Analysis of Illness Perception and Acceptance.

authors

- Liu Y
- Shen C
- Huang Y
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- Zhang X
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- Zhou J
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journal: Nursing open

journalAbbr: Nurs Open

year: 2026

pubdate: 2026 Aug

volume: 13

issue: 8

pages: e70684

abstract: AIMS: This study examined the effect of frailty on social isolation among elderly prostate cancer patients, focusing on the chain mediating roles of illness perception and acceptance of illness. Additionally, a network analysis was conducted to explore the internal interconnections between illness perception and acceptance of illness. DESIGN: Descriptive quantitative study. METHODS: A total of 288 elderly prostate cancer patients were recruited from a major public hospital in Wenzhou. The study investigated participants' baseline information, frailty, illness perception, acceptance of illness, and social isolation. Chain mediation analysis was conducted using SPSS 26.0 and the PROCESS Macro, whereas partial correlation network analysis was performed in R. RESULTS: Frailty, illness perception, and acceptance of illness were all significantly associated with social isolation. The link between frailty and social isolation was sequentially mediated by illness perception and acceptance of illness, explaining 61% of the total variance. Among these factors, acceptance of illness exerted the strongest effect. Network analysis further indicated the interrelationship between illness perception and acceptance of illness. CONCLUSION: This study highlights the importance of addressing frailty, illness perception, and acceptance of illness in clinical nursing practice, which may help reduce social isolation among prostate cancer patients. IMPLICATIONS FOR THE PROFESSION OR PATIENT CARE: Recognizing frailty, alleviating negative illness perceptions, and enhancing illness acceptance may help prevent social isolation related to illness. IMPACT: This study provides a theoretical basis for elucidating the mechanisms of social isolation and provides precise targets for clinical intervention. Nurses can integrate various treatment approaches from these views to enhance the health and well-being of elderly prostate cancer patients. REPORTING METHOD: The STROBE guidelines were followed. PATIENT OR PUBLIC CONTRIBUTION: Patients contributed by completing the questionnaire, ensuring the accuracy and completeness of the information with assistance from the research team.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13437946/

Record 60

pmcid: PMC13631031

pmid: 42414646

doi: 10.1007/s00266-026-06027-3

title: From Etiology to Aesthetic Surgery: Mapping Knowledge Domains and Emerging Trends in Gynecomastia Research: 35-Year Bibliometric Analysis.

authors

- Deng K
- Liu Y
- Tang Y
- Xie N

journal: Aesthetic plastic surgery

journalAbbr: Aesthetic Plast Surg

year: 2026

pubdate: 2026 Sep

volume: 50

issue: 17

pages: 7109-7123

abstract: BACKGROUND: Gynecomastia is a benign proliferation of glandular tissue in the male breast, with incidence rate of 30-60% among adolescents. Its etiology involves physiological hormonal fluctuations, pathological conditions, and pharmacologic factors. It causes psychological burdens like body dysmorphic disorder and anxiety. Traditional reviews can't synthesize numerous fragmented evidence, making objective and quantitative analysis needed. METHOD: The Web of Science Core Collection (SCIE) was data source to search literature about "gynecomastia" from 1990 to December 2024. After PRISMA process and triple-blind screening by three plastic surgery doctors, 863 English original articles were included. Software like CiteSpace, VOSviewer, and Excel were used for bibliometric and visual analysis. RESULT: Research showed three-stage stepwise growth trend. The United States was core of research (203 publications, betweenness centrality of 0.37). High-productivity institutions were mostly comprehensive universities and specialized hospitals. Author collaboration was decentralized. Core journals focused on plastic surgery and endocrinology fields. 12 keyword clusters were identified. Clusters with latest average years were #0 chest remodeling, #8 androgen receptor, and #9 dietary supplements. The ongoing hot issues include "risk factors," "minimally invasive surgery," "endoscopic treatment," "hair removal," "crystallized phenol," "quality of life," "explanation minimally invasive treatment," and "efficacy." CONCLUSION: This study visually presents research landscape and developmental trends in the gynecomastia field since 1990. Future research should focus on cases about elderly population, establish multidisciplinary teams (MDT), strengthen real-world studies, resolve conflicts between individual experiences, esthetics, and traditional social gender norms, and explore new technologies' application like 3D printing and AI-assisted analysis. LEVEL OF EVIDENCE III: This journal requires that authors assign a level of evidence to each article. For a full description of these Evidence-Based Medicine ratings, please refer to the Table of Contents or the online Instructions to Authors. www.springer.com/00266 .

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13631031/

Record 61

pmcid: PMC13549563

pmid: 42708116

doi: 10.1097/GOX.0000000000008125

title: Galactocele Rupture Causing Milk Contamination of a Prepectoral Implant Pocket: Diagnostic Pitfall and Implant Salvage.

authors

- Ongkasuwan P

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2026

pubdate: 2026 Sep

volume: 14

issue: 9

pages: e8125

abstract: Galactocele is a benign milk retention cyst that may develop during or after lactation. In patients with breast implants, ruptured galactoceles can cause peri-implant fluid collections that mimic implant rupture or infection, creating a diagnostic challenge. This report describes a 37-year-old Asian woman, G1P1, with bilateral prepectoral smooth-surface silicone breast implants who presented 2 months postpartum with progressive left breast swelling that was initially diagnosed as mastitis. After noticing a palpable mass, the patient performed vigorous self-massage and increased breast pumping in an attempt to relieve presumed milk stasis, which precipitated acute breast enlargement. Despite antibiotic therapy, symptoms worsened, and evaluation by a radiologist and a general surgeon resulted in a diagnosis of implant rupture with infection and abscess; galactocele rupture was not considered. Surgical exploration revealed an intact implant with milk contamination of the implant pocket due to galactocele rupture and duct-capsule communication. No mature capsule was

identified; the pocket was lined with granulation tissue. Fluid analysis demonstrated elevated triglyceride levels consistent with milk content, whereas bacterial cultures remained negative. Management included closure of the ductal communication, sequential antiseptic irrigation, drain placement, and lactation suppression with cabergoline. The implant was successfully salvaged, and the patient remained asymptomatic at 3-month follow-up without capsular contracture. Galactocele rupture should be considered in the differential diagnosis of peri-implant fluid collections in postpartum patients with breast implants. Vigorous manipulation over a palpable mass should be avoided, and timely lactation suppression is critical for implant salvage regardless of closure integrity.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13549563/

Record 62

pmcid: PMC13043454

pmid: 41938278

doi: 10.1016/j.aed.2025.12.006

title: Galactorrhea and Pituitary Microadenoma in an 18-Year-Old Transfeminine Youth: A Case Report.

authors

- Bongsebandhu-Phubhakdi C
- Jakchairoongruang K
- Poonmaksatit S

journal: AACE endocrinology and diabetes

journalAbbr: AACE Endocrinol Diabetes

year: 2026

pubdate: 2026 Mar-Apr

volume: 13

issue: 2

pages: 313-317

abstract: BACKGROUND/OBJECTIVE: Gender-affirming hormone therapy (GAHT) improves psychological well-being in transfeminine individuals but may cause endocrine alterations, including hyperprolactinemia and, rarely, pituitary adenomas. We report the youngest known case of galactorrhea and pituitary microadenoma associated with unsupervised GAHT, highlighting multifactorial contributing factors. CASE REPORT: An 18-year-old transfeminine youth presented with galactorrhea while taking self-administered GAHT without medical supervision. Laboratory testing showed elevated prolactin, and magnetic resonance imaging revealed a pituitary microadenoma. Contributing factors included high-dose estradiol, cyproterone acetate, concomitant antiretroviral and antidepressant therapy, and frequent breast massage. DISCUSSION: This case highlights the endocrine vulnerability of adolescents using unsupervised GAHT. The combined effects of estrogen excess, antiandrogen therapy, serotonergic and dopaminergic modulation, and mechanical breast stimulation likely contributed to hyperprolactinemia. Limited pediatric data and lack of structured hormonal monitoring delayed diagnosis and optimal management. Multidisciplinary care integrating endocrine follow-up and mental health support is essential to ensure treatment safety. CONCLUSION: Unsupervised GAHT in adolescents can lead to significant endocrine complications through multifactorial mechanisms. Early recognition, structured monitoring, and coordinated multidisciplinary care are crucial to balance gender-affirming goals with patient safety.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13043454/

Record 63

pmcid: PMC13268603

pmid: 42306095

doi: 10.3389/fgene.2026.1827876

title: Genetic analysis of three familial cases of pure terminal 19p13.3 duplication caused by maternal balanced translocation t(19;21) (p13.3;p12).

authors

- Chen JY
- Cai MJ
- Chen XL
- Ge YS

journal: Frontiers in genetics

journalAbbr: Front Genet

year: 2026

pubdate: 2026

volume: 17

issue:

pages: 1827876

abstract: INTRODUCTION: This study presents a genetic analysis of three-generation family exhibiting terminal 19p13.3 duplication resulting from a maternal balanced translocation, t (19; 21) (p13.3; p12). Additionally, we reviewed previously reported cases with similar aberrations. METHODS: Peripheral blood or amniotic fluid samples from six family members were comprehensively analyzed using G-banding, N-banding, fluorescence in situ hybridization (FISH), and copy number variation sequencing (CNV-seq). Furthermore, a genomic mapping analysis was performed on 23 cases of terminal 19p13.3 duplication, combining our findings with those from previously reported cases. RESULTS: The results demonstrated that the maternal balanced translocation resulted in three offspring inheriting the 19p13.3 terminal duplication. The proband presented with typical clinical features such as intrauterine growth restriction, microcephaly, intellectual disability, developmental delay, and facial abnormalities, in addition to precocious puberty, autism spectrum disorder (ASD) features, and a shortened lingual frenulum. The precocious puberty phenotype is postulated to be associated with the KISS1R gene within the duplicated region. Genomic mapping identified a minimal overlapping region (MOR) of approximately 313 kb (chr19:3,223,850-3,536,224), encompassing four OMIM genes (CELF5, NFIC, DOHH, FZR1) and two protein-coding genes (SMIM24, SMIM44). CONCLUSION: This study clarifies the genetic mechanism of pure terminal 19p13.3 duplication in offspring resulting from a parental balanced translocation involving D/G group chromosomes. It also defines a novel critical region based on case samples with 19p13.3 terminal duplication, providing new insights into the genotype-phenotype correlations associated with terminal pure 19p13.3 duplications.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13268603/

Record 64

pmcid: PMC13652467

pmid: 42857398

doi: 10.1097/RC9.0000000000000977

title: Gestational gigantomastia in the early postpartum period presenting with severe refractory pain: a case report.

authors

- Assefa EA
- Dinagde TA
- Yusuf L
- Shumiye YG
- Nuru EM

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2026

pubdate: 2026 Oct

volume: 138

issue: 10

pages: 4477-4482

abstract: INTRODUCTION: Gestational gigantomastia (GG) is a rare, benign breast disorder characterized by rapid, excessive, and disproportionate breast enlargement during pregnancy. Affected patients may develop severe breast pain, skin breakdown, secondary infection, and musculoskeletal complications, resulting in significant functional impairment and psychosocial distress. CASE PRESENTATION: We report the case of a 24-year-old woman (G2P1A1) who presented on the second postpartum day with a 5-month history of rapidly progressive bilateral breast enlargement and severe, disabling breast pain of 2 days' duration. Despite treatment with bromocriptine and multimodal analgesia, her pain remained refractory, necessitating emergent bilateral simple mastectomy. Histopathological examination confirmed GG, and the patient had an uneventful postoperative recovery. DISCUSSION: GG is an uncommon clinical entity, with an estimated incidence ranging from 1 in 28 000 to 1 in 100 000 pregnancies. Although hormonal hypersensitivity is presumed to play a role, its etiology remains poorly understood. Medical management is generally recommended as first-line therapy; however, complete regression is uncommon, and the clinical response is often limited. Consequently, surgical management remains the definitive treatment option in many cases, particularly when complications arise or conservative therapy fails. CONCLUSION: Early postpartum GG with severe refractory pain requiring emergent bilateral mastectomy is exceedingly rare. Bilateral mastectomy with planned reconstruction may be an appropriate definitive treatment option when conservative measures fail, particularly for patients wishing to reduce the risk of recurrence during future pregnancies. Furthermore, this approach avoids the potential risk of life-threatening hemorrhage associated with the alternative surgical option of reduction mammoplasty.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13652467/

Record 65

pmcid: PMC13377018

pmid: 42491248

doi: 10.70352/scrj.cr.26-0307

title: Giant Benign Phyllodes Tumor Presenting with Skin Ulceration: A Case Report and Review of the Literature.

authors

- Kawabata M
- Tanaka K
- Tohyama S
- Akima R
- Yamamoto R
- Miyashita M
- Kato H
- Sakuraba S
- Wada R
- Ito T
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journal: Surgical case reports

journalAbbr: Surg Case Rep

year: 2026

pubdate: 2026

volume: 12

issue: 1

pages:

abstract: INTRODUCTION: Phyllodes tumors of the breast are rare fibroepithelial neoplasms accounting for 0.3%-1.0% of all primary breast tumors. They are classified into benign, borderline, and malignant subtypes, with borderline and malignant tumors associated with an increased risk of local recurrence and distant metastasis. Giant phyllodes tumors, defined as tumors larger than 10 cm in diameter, are uncommon, and reports of tumors exceeding 20 cm are extremely rare. We report a case of a rapidly growing giant benign phyllodes tumor with skin ulceration, along with a review of the relevant literature. CASE PRESENTATION: A 48-year-old Asian woman presented with rapid enlargement of the left breast and active bleeding from an ulcerated mass. Clinical findings and imaging studies revealed a well-circumscribed giant tumor measuring 24 cm that occupied the entire left breast, without evidence of distant metastasis. Core needle biopsy suggested a fibroepithelial tumor, and a benign phyllodes tumor was most strongly considered. A left mastectomy was performed. Skin grafting had been considered preoperatively, as the skin defect would be large. However, as sufficient skin flaps were obtained intraoperatively, due to the skin being stretched by the tumor, only mastectomy was done without a reconstructive procedure. Histopathological examination confirmed a benign phyllodes tumor with negative surgical margins. The postoperative course was uneventful. CONCLUSIONS: We experienced a rare case of a giant breast tumor and tumor resection. Although rapid breast tumor growth and skin breakdown suggested malignancy, the pathological diagnosis was a benign phyllodes tumor. This highlights the importance of considering phyllodes tumors in the differential diagnosis of rapidly enlarging breast masses and the need for timely surgical intervention. This case is particularly noteworthy because it demonstrates an unusual combination of clinical features-giant tumor size, rapid growth, and skin ulceration-that are generally associated with malignant disease, despite the final diagnosis being benign.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13377018/

Record 66

pmcid: PMC13276934

pmid: 42026594

doi: 10.1186/s12887-026-06898-0

title: Growth hormone therapy in children with temple syndrome: the first retrospective study in China.

authors

- Wang X
- Chen R
- Su Z
- Fan X
- Shen L
- Song F
- Liu Z
- Gong C
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journal: BMC pediatrics

journalAbbr: BMC Pediatr

year: 2026

pubdate: 2026 Apr 23

volume: 26

issue: 1

pages:

abstract: BACKGROUND: Temple syndrome (TS14) is a rare imprinting disorder caused by dysregulation of imprinted genes in the 14q32 region. Although several cohorts have been reported, pediatric data from China are still lacking. This study aimed to characterize the clinical features in Chinese children with TS14 and evaluate their response to recombinant human growth hormone (rhGH), including outcomes associated with initiating treatment before two years of age. METHODS: A multicenter retrospective review was conducted involving seven children diagnosed with TS14 across six pediatric centers in China. Clinical characteristics, growth parameters, genetic testing, and treatment outcomes were extracted from medical records. Molecular diagnosis was established by methylation-specific assays, with additional molecular-genetic testing performed as indicated. Changes in height standard deviation score (SDS) were analyzed descriptively. RESULTS: Seven patients (1 male, 6 females) were included, with a median age at diagnosis of 2.0 (IQR: 0.67–7.33) years. Maternal UPD(14) was confirmed in one case and strongly suspected in three cases, whereas in the remaining three cases the underlying mechanism could not be determined. Most patients were small for gestational age (71.4%) and all exhibited growth retardation, delayed language development, dysmorphic features, with feeding difficulties (85.7%), and hypotonia (71.4%). All three older children subsequently developed central precocious puberty. Six patients who were initiated on rhGH therapy before puberty (median age at initiation, 2.42 years), achieved a mean height SDS increase of 1.19 after one year. Among the three patients treated before age two, the mean height SDS gain was 1.15, with caregiver-reported improvements in motor and language development, and no adverse events reported. CONCLUSIONS: This inaugural TS14 cohort from China shows clinical features largely consistent with those reported in international cohorts, with a high frequency of precocious puberty among older patients. rhGH therapy yielded substantial short-term gains in height, and initiation of treatment before age two was accompanied by caregiver-reported developmental improvements, although standardized developmental assessments were not available. Early genetic diagnosis and timely intervention are essential, while long-term outcomes require further study.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13276934/

Record 67

pmcid: PMC13302684

pmid: 42355536

doi: 10.3390/life16061010

title: Gynecomastia: Etiological Analysis Beyond Hormonal Imbalance.

authors

- Kim JH
- Song M
- Lee K
- Lee MH
- Song YS

journal: Life (Basel, Switzerland)

journalAbbr: Life (Basel)

year: 2026

pubdate: 2026 Jun 16

volume: 16

issue: 6

pages:

abstract: Background: Gynecomastia is a benign enlargement of the male breast with multiple causes. This retrospective study evaluated whether serum estradiol (E2), testosterone (T), or the E2/T ratio differed between patients and controls, while explicitly accounting for the limited hormonal subset and lack of statistical significance. Methods: We retrospectively reviewed 68 men with benign gynecomastia (1996-2000) and 10 healthy controls. Analyses were conducted in two predefined populations: a full etiologic cohort (n = 68) and a hormonal-analysis subset (37 patients and 10 controls). Serum E2 and T were measured in 37 patients and all controls, and the E2/T ratio was calculated. The E2/T ratio was analyzed primarily as a continuous variable rather than relying solely on a control-derived threshold. Results: Gynecomastia was most frequent in adult and middle-aged men. Idiopathic cases were most common, followed by drug-related, hormonal, and disease-related causes. The mean E2/T ratio was higher in patients (12.0 ± 1.8) than controls (7.1 ± 0.5), but this difference did not reach statistical significance (E2: p = 0.21; T: p = 0.34; E2/T: p = 0.07). Among tested patients, 13.5% had low T, 24.3% high E2, and 43.2% elevated E2/T ratios. Liver disease and drugs such as H 2 blockers and psychoactive agents contributed in some cases, while 50% were idiopathic. Conclusions: Because hormone comparisons were non-significant and based on a limited subset, these data do not support an independent association between gynecomastia and circulating E2, T, or the E2/T ratio. Clinical evaluation should consider age, comorbidities, and medications in addition to hormonal factors.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13302684/

Record 68

pmcid: PMC13490620

pmid:

doi: 10.1111/jocd.71147

title: Hydrodynamic-Assisted Liposuction With Stab-Flatten Technique Versus Traditional Excision for Gynecomastia: A Retrospective Cohort Study.

authors

- Zhang J
- Pei J
- Liu C
- Ding J
- Cui J
- Yan B
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journal: Journal of Cosmetic Dermatology

journalAbbr: J Cosmet Dermatol

year: 2026

pubdate: 2026 Aug

volume: 25

issue: 8

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13490620/

Record 69

pmcid: PMC12744928

pmid: 41467070

doi: 10.3892/ol.2025.15428

title: Identification of non-oncology drugs with activity against inflammatory and triple-negative breast cancer cell lines.

authors

- Ahmad S
- Kandil S
- Williams KP
- Scott JE

journal: Oncology letters

journalAbbr: Oncol Lett

year: 2026

pubdate: 2026 Feb

volume: 31

issue: 2

pages: 75

abstract: Inflammatory breast cancer (IBC) and triple-negative breast cancer (TNBC) are subtypes of breast cancer with the lowest 5-year survival rates. One potential approach to increase survival rates for these cancers may be to identify tolerable and inexpensive daily maintenance therapies that could be administered in between standard of care treatments. The present study aimed to identify non-oncology drugs (NODs) that show activity against IBC/TNBC that could be repurposed for this use. Thus, a collection of NODs were screened and curated from a published database using cell line SUM-149, which is both an IBC and TNBC cell line (TN-IBC). In a parallel screening approach, a subset of these NODs were tested for activity against two other TNBC cell lines. Together, 15 drugs with activity against this IBC cell line were identified and grown as spheroids. A number of these drugs have not been previously identified as having activity in an IBC cell line. In addition, eight of these were understudied for TNBC and, therefore, they were tested against three TNBC cell lines grown as spheroids, resulting in variable breadth of activities. Drug potency (GI 50) was also assessed relative to their C max reported in human studies. Eltrombopag, a thrombocytopenia drug, was active against SUM-149 and two other TNBC cell lines with a GI 50 :C max ratio of ~1. Papaverine, a vasodilator, was also active against SUM-149 and one other TNBC cell line with a GI 50 :C max ratio of ~2. Mozavaptan, a hyponatremia drug, inhibited proliferation in 4 out of 5 cell lines (all except SUM-149) with GI 50 =2.7-7.6 µM (C max unknown). Spironolactone (GI 50 :C max =10) had anti-proliferative activity only against SUM-149, suggesting a unique vulnerability. Overall, our data indicated that eltrombopag, papaverine and other identified NODs may have potential for being repurposed for TN-IBC and TNBC in general.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12744928/

Record 70

pmcid: PMC12874181

pmid: 41658818

doi: 10.7759/cureus.100810

title: Idiopathic Recurrent Unilateral Pneumomastia: Diagnostic Challenges and Therapeutic Considerations With a Rare Case of Mammary Emphysema.

authors

- Albalawi H
- Alqarni SM

- Alsati WA
- Alamassi MY
- Elkhouly AG

journal: Cureus

journalAbbr: Cureus

year: 2026

pubdate: 2026 Jan

volume: 18

issue: 1

pages: e100810

abstract: Pneumomastia is a rare condition characterized by air within the breast tissue. It is typically caused by trauma, iatrogenic factors, or infections. We report a 22-year-old female patient with a 20-day history of progressive, painless unilateral left breast swelling and subcutaneous emphysema following a prior spontaneously resolved episode in the contralateral breast. Imaging revealed extensive extrapulmonary air within the left breast with subcutaneous emphysema. After exclusion of life-threatening causes, including pneumothorax and esophageal perforation, ultrasound-guided needle aspiration was performed, providing symptomatic relief but complicated by post-procedural infective mastitis. While most cases resolve spontaneously, invasive interventions should be guided by symptom severity. At the 12-month follow-up, the patient remained asymptomatic with no recurrence. A review of the literature indicates that idiopathic recurrent pneumomastia is an exceptionally uncommon presentation with scarce documented cases. This case highlights diagnostic challenges and outlines management strategies in idiopathic recurrent pneumomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12874181/

Record 71

pmcid: PMC13094907

pmid: 42016419

doi: 10.4103/jehp.jehp_354_25

title: Influence of biopsychosocial factors on pregnant women during perinatal period.

authors

- Fadhil SN

journal: Journal of education and health promotion

journalAbbr: J Educ Health Promot

year: 2026

pubdate: 2026

volume: 15

issue:

pages: 99

abstract: BACKGROUND: The perinatal period is shaped by biopsychosocial factors, including biological changes, psychological stressors, and social determinants like socioeconomic status and support systems. These factors collectively impact maternal and neonatal health. While individual aspects have been studied, research on their combined effects, particularly in local contexts, remains limited. This study aimed to determine the influence of biopsychosocial factors on pregnant women's well-being during the perinatal period. This research seeks to fill a gap in current knowledge and provide insights that can improve healthcare strategies and maternal-child health outcomes. MATERIALS AND METHODS: A descriptive study was conducted from February to April with 100 pregnant women attending obstetrics clinics in Baghdad, Iraq. Data was collected

through face-to-face interviews using a structured questionnaire covering demographic, reproductive, biological, psychological, and social factors. Statistical analysis was performed using SPSS version 26. RESULTS: Social factors (age 25-29, education, social support, and planned pregnancy) significantly affected pregnant women. Most participants had physical issues, but these did not significantly impact adverse maternal outcomes, except for pregnancy weight gain (P = 0.000). Psychological factors showed moderate concern (64%) and were highly associated with adverse outcomes (P = 0.000). CONCLUSION: This study underscores the importance of considering biopsychosocial factors in perinatal care. The findings highlight the need for targeted healthcare interventions that address these factors to improve maternal and neonatal health outcomes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13094907/

Record 72

pmcid: PMC13023227

pmid: 41871008

doi: 10.12659/AJCR.947844

title: Isolated Premature Menarche in a 17-Month-Old: A Case Report.

authors

- Pelitawati DR
- Susilo AFP
- Rachmawati A
- Permadi W
- Adriansyah PNA
- Hasanah NC

journal: The American journal of case reports

journalAbbr: Am J Case Rep

year: 2026

pubdate: 2026 Mar 23

volume: 27

issue:

pages: e947844

abstract: BACKGROUND Vaginal bleeding in prepubertal girls is an uncommon finding and should always prompt thorough evaluation. Puberty is a complex physiological process resulting from maturation of the hypothalamic-pituitary-gonadal axis. Precocious puberty is defined as the early onset of secondary sexual characteristics before the age of 8 years in girls. The incidence ranges from 1 in 5000 to 1 in 10 000 girls worldwide. In contrast, isolated premature menarche is a rare and benign condition characterized by vaginal bleeding without other signs of pubertal activation and remains a diagnosis of exclusion. CASE REPORT We report a 17-month-old female infant who presented with recurrent vaginal bleeding every month over the 3 months prior to admission. Physical examination showed normal external genitalia, with no signs of inflammation, trauma, foreign body, or sexual abuse. There were no secondary sexual characteristics, and growth parameters were appropriate for age. Laboratory examination revealed prepubertal basal gonadotropin levels, normal hormonal levels, normal thyroid function tests, and bone age consistent with chronological age. Platelet function testing showed hyperaggregation, considered a transient and incidental finding. Pelvic ultrasonography demonstrated the size of the uterus was consistent with the age of puberty. After exclusion of local, endocrine, and systemic causes, a diagnosis of isolated premature menarche was established. CONCLUSIONS Menstrual-like vaginal bleeding in prepubertal girls requires systematic and comprehensive examination to differentiate isolated premature menarche from other diseases. Recognition of this benign entity is important to avoid unnecessary interventions and ascertain appropriate follow-up.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13023227/

Record 73

pmcid: PMC13180294

pmid: 42153050

doi: 10.7759/cureus.107219

title: Male Tuberous Breast: A Rare Variant of Gynecomastia.

authors

- Ankiz A
- El Kadiri S
- Arreyouchi D
- Oufkir AA

journal: Cureus

journalAbbr: Cureus

year: 2026

pubdate: 2026 Apr

volume: 18

issue: 4

pages: e107219

abstract: Tuberous breast deformity is well described in females but remains rare in males, where it represents an uncommon morphological variant of gynecomastia. It is characterized by a constricted breast base, enlarged nipple-areolar complex (NAC), inframammary fold indentation, and apparent parenchymal herniation into the areola. We report the case of a 17-year-old male presenting with progressive bilateral breast enlargement since the age of 11, with marked worsening at 16 years and significant psychosocial distress. Clinical examination revealed bilateral glandular gynecomastia with enlarged NACs, mild asymmetry, and global base constriction consistent with a severe tuberous deformity. Ultrasonography showed homogeneous glandular hypertrophy, and endocrine evaluation was normal. Surgical correction using glandular transection and release of constricting elements resulted in satisfactory aesthetic outcomes without complications. Male tuberous breast deformity is an underrecognized entity requiring tailored surgical management addressing both glandular excess and base constriction.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13180294/

Record 74

pmcid: PMC13390745

pmid: 42488929

doi: 10.1093/jscr/rjag478

title: Malignant phyllodes tumor arising from a previously biopsy-proven fibroadenoma: a case report.

authors

- Venkatesan A
- Brown A
- Katragadda N
- Light K
- Ronaghan C

journal: Journal of surgical case reports

journalAbbr: J Surg Case Rep

year: 2026

pubdate: 2026 Jul

volume: 2026

issue: 7

pages: rjag478

abstract: Fibroadenomas are common benign lesions that are typically managed conservatively. However, transformation into malignant phyllodes tumor is a rare but critical progression that demands a shift from surveillance to surgical intervention. We present the case of a patient who underwent multiple core-needle biopsies over several years, all confirming a fibroadenoma, yet the mass demonstrated progressive growth, ultimately exceeding 7 cm. Ultimately, surgical excision revealed a malignant phyllodes tumor characterized by stromal overgrowth, cellular atypia, increased mitotic activity, and infiltrative margins. This case highlights the diagnostic limitations of core-needle biopsy in fibroepithelial lesions. Although the vast majority of fibroadenomas remain benign, a minority may undergo malignant transformation. Physicians must maintain a high degree of clinical suspicion and closely monitor longstanding lesions for any signs of growth or change. Early distinction between fibroadenomas and phyllodes tumors is essential, as management strategies differ substantially.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13390745/

Record 75

pmcid: PMC13627163

pmid: 42258023

doi: 10.1007/s40618-026-02915-2

title: Management of gynecomastia in adolescence and adults: the clinical practice guidelines from the Italian Society of Andrology and Sexual Medicine (SIAMS).

authors

- Pozza C
- Selice R
- Barbonetti A
- Hasenmajer V
- Lotti F
- Menafra D
- Pasquali D
- Sansone A
- Isidori AM
- Rochira V

journal: Journal of endocrinological investigation

journalAbbr: J Endocrinol Invest

year: 2026

pubdate: 2026 Oct

volume: 49

issue: 10

pages: 2583-2610

abstract: PURPOSE: Gynecomastia, defined as the benign proliferation of male breast glandular tissue, is a common condition in both adolescence and adulthood, often associated with physical discomfort and

psychological distress. Despite its high prevalence, clinical management is frequently inconsistent and lacks standardized, evidence-based recommendations. METHODS: The Italian Society of Andrology and Sexual Medicine (SIAMS) convened a multidisciplinary expert panel to develop updated clinical practice guidelines for the evaluation and management of gynecomastia. Recommendations were formulated based on a systematic review of the available literature and developed through expert consensus according to the GRADE (Grading of Recommendations, Assessment, Development, and Evaluation) system. The document was critically reviewed and approved by the SIAMS Executive Committee. RESULTS: The guidelines address physiological, pharmacological, pathological, congenital, and genetic forms of gynecomastia, providing a structured diagnostic and therapeutic approach. Clinical assessment, including detailed history and examination, is the cornerstone of diagnosis, and should be complemented by targeted hormonal and imaging investigations when appropriate. Observation is recommended for physiological or recent-onset cases. Pharmacological therapy may be considered in selected idiopathic or painful forms, whereas testosterone replacement should be restricted to men with proven hypogonadism. Surgery is indicated for long-standing fibrotic gynecomastia or significant psychological distress. Psychological support is also advised, especially for adolescents. CONCLUSIONS: These SIAMS Guidelines provide an evidence-based, standardized framework for the diagnosis and management of gynecomastia, promoting individualized, multidisciplinary care and identifying priorities for future research.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13627163/

Record 76

pmcid: PMC13557779

pmid: 42719506

doi: 10.7759/cureus.114300

title: Marked Unilateral Breast Enlargement Caused by a Fibroepithelial Lesion Suspicious for Phyllodes Tumor in the Modern Screening Era: A Case Report and Literature Review.

authors

- Izadian Bidgoli A
- Joseph R
- Yazdanpanah S
- Gonzalez G

journal: Cureus

journalAbbr: Cureus

year: 2026

pubdate: 2026 Aug

volume: 18

issue: 8

pages: e114300

abstract: Markedly enlarged fibroepithelial breast lesions have become increasingly uncommon in the modern era of routine breast imaging and earlier diagnostic evaluation. We report the case of a 52-year-old woman who presented with progressive unilateral right breast enlargement and pain after delaying evaluation despite previously abnormal breast imaging. Clinical examination demonstrated marked right breast enlargement without overt inflammatory or infectious changes. Ultrasound-guided core needle biopsy revealed a fibroepithelial lesion with benign phyllodes tumor included in the differential diagnosis; however, definitive histologic classification required excisional pathology. Given the extensive breast involvement and limited feasibility of breast-conserving surgery, the case was reviewed by a multidisciplinary team, and total mastectomy was recommended. At the time of manuscript preparation, the patient was still considering surgical management, and definitive excisional pathology and long-term follow-up were unavailable. This case highlights the uncommon contemporary presentation of a markedly enlarged fibroepithelial breast lesion despite advances

in breast screening and emphasizes the diagnostic limitations of core needle biopsy, the importance of multidisciplinary evaluation, and the consequences of delayed medical assessment on subsequent management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13557779/

Record 77

pmcid: PMC13537616

pmid: 42688984

doi: 10.7759/cureus.113887

title: Massive Bilateral Breast Hypertrophy in Pregnancy Due to Gestational Gigantomastia: A Case Report and Review of the Literature.

authors

- El Mangoub F
- Benlghazi A
- Yassine B
- Elhassani MM
- Jaouad K

journal: Cureus

journalAbbr: Cureus

year: 2026

pubdate: 2026 Aug

volume: 18

issue: 8

pages: e113887

abstract: Gestational gigantomastia is a very rare disorder characterised by massive, bilateral breast enlargement during pregnancy. Clinically, it is characterised by rapid breast enlargement associated with severe mastodynia, back and shoulder pain, and erythema of the skin. In its most severe forms, it can lead to ulceration, necrosis, haemorrhage, or septicaemia. The underlying pathophysiology is not completely understood, but is thought to be multifactorial, involving hormonal hypersensitivity and autoimmune mechanisms. Histopathological confirmation is necessary for definitive diagnosis because its clinical presentation can closely mimic that of inflammatory breast carcinoma or other malignant neoplasms. We report a case of a 28-year-old primigravida who presented at 24 weeks of gestation with progressive bilateral breast enlargement that reached its maximal extent at 36 weeks and caused significant functional impairment. On clinical examination, there was symmetrical breast hypertrophy with diffuse erythema and a marked peau d'orange appearance. Breast ultrasound revealed glandular hypertrophy with ductal ectasia and marked oedema. No suspicious focal lesions were seen. Hormonal and immunological workup was normal, and skin and glandular biopsy showed benign hyperplasia without atypia. Throughout the pregnancy, conservative management with analgesics and postural support was continued until an uncomplicated vaginal delivery at 39 weeks of gestation. Cabergoline was then administered to suppress lactation. Given the absence of spontaneous regression of the breasts, a multidisciplinary decision was taken to perform a bilateral reduction mammoplasty at 12 months postpartum. This case demonstrates the phenotypic variability of gestational gigantomastia that may present without any preceding breast pathology. In severe cases, surgical treatment (reduction mammoplasty or bilateral mastectomy with delayed reconstruction) is the mainstay of treatment, and bromocriptine is the best-established medical alternative, but with variable success. Coordinated multidisciplinary care is essential to optimise patient outcomes and minimise life-threatening complications.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13537616/

Record 78

pmcid: PMC12676515

pmid: 41358270

doi: 10.3892/ol.2025.15398

title: Massive breast fat necrosis with invasive breast cancer: A case report.

authors

- Zhang A
- Song L
- Yang X
- Chen X
- Tian B
- Li Y
- Jiang G
- Jiang L

journal: Oncology letters

journalAbbr: Oncol Lett

year: 2026

pubdate: 2026 Feb

volume: 31

issue: 2

pages: 45

abstract: Breast fat necrosis is a relatively uncommon benign breast condition, while invasive breast cancer is one of the most common types of breast cancer. Coexistence of massive breast fat necrosis with invasive breast cancer is relatively uncommon. The present study reported the case of a 74-year-old female patient who was diagnosed with breast cancer based on mammography findings; breast MRI indicated fat necrosis; subsequent surgical resection confirmed the coexistence of breast fat necrosis with invasive breast cancer using histopathological and immunohistochemical analysis. The relatively uncommon combination of these two conditions poses notable diagnostic challenges for radiologists. The present study reported relevant imaging findings to improve the understanding of radiologists on this disease, highlighting the clinical and radiological features.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12676515/

Record 79

pmcid: PMC12884414

pmid: 41669573

doi: 10.7759/cureus.101120

title: McCune-Albright Syndrome: A Case Series.

authors

- Agarwal V
- Das S
- Chappalagavi AS
- Meher D
- Sahoo D

journal: Cureus

journalAbbr: Cureus

year: 2026

pubdate: 2026 Jan

volume: 18

issue: 1

pages: e101120

abstract: McCune- Albright syndrome (MAS) is a rare disorder characterized by postzygotic somatic activation mutation of GNAS, leading to constitutively active Gsα and ligand-independent signaling of the Gs-coupled protein receptor. Polyostotic fibrous dysplasia, café-au-lait macules, and endocrine hyperfunction, commonly gonadotropin-independent precocious puberty (GIPP), are the three clinical hallmarks of this condition. Mosaicism results in a wide range of clinical presentations. In this case series, we present three cases of MAS with varied presentations. Case 1 describes a 9-year-old boy with recurrent fractures and craniofacial fibrous dysplasia. Case 2 is an 8-year-old girl with early menarche and GIPP. Case 3 details a 7-year-old girl with unilateral early breast development progressing from GIPP to gonadotropin-dependent precocious puberty (GDPP). As it is a heterogeneous disorder, it requires individualized management. Prompt recognition of clinical features can aid early diagnosis and intervention to mitigate disease progression.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12884414/

Record 80

pmcid: PMC12729506

pmid: 41439563

doi: 10.1111/jocd.70628

title: Medicinal Plants for Acne Vulgaris: An Evidence-Based Review of Treatments Promoted by Social Media.

authors

- Parvizi MM
- Saki N
- Rostami Ghotbabadi Z
- Kamali M
- Salmanpour N
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journal: Journal of cosmetic dermatology

journalAbbr: J Cosmet Dermatol

year: 2026

pubdate: 2026 Jan

volume: 25

issue: 1

pages: e70628

abstract: BACKGROUND: Acne vulgaris refers to a chronic inflammatory state of the pilosebaceous follicles that affects the majority of adolescents. Treatments encompass topical agents and systemic therapies. Nowadays, we encounter a growing tendency to use herbal remedies, which raises concerns about misinformation disseminated by digital platforms. AIM: This narrative review investigated herbal acne treatments recommended by virtual platforms (WhatsApp, Instagram, and Telegram), and gleaned medicinal plants from authoritative references. METHODS: A literature search across international and Persian databases, including PubMed and Scopus, was implemented employing keywords and Boolean operators. Data synthesis highlighted evidence

gaps and inconsistencies. RESULTS: The review found different medicinal plants holding potential acne treatment properties, including tea tree oil, lavender, licorice, turmeric, and heartsease among others. Some demonstrated antibacterial, anti-inflammatory, or sebum-reducing effects in vitro or clinically. Others lacked strong evidence or clinical validation. Furthermore, combinations like tea tree and lavender oils showed lesion-decreasing effects. However, gaps remain in clinical research for a number of plants traditionally claimed to treat acne vulgaris. CONCLUSION: While digital platforms play an important role in spreading health-related information, they have promoted several remedies for acne vulgaris that have not been scientifically verified. Hence, cyberspace is not currently deemed a dependable source of treatment information for nonmedical experts.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12729506/

Record 81

pmcid: PMC13381241

pmid: 42518806

doi: 10.3389/fonc.2026.1892580

title: Metastatic malignant melanoma presenting as bilateral acute mastitis in a lactating woman: a case report and literature review.

authors

- Wang Z
- Zeng S
- Zhou Q
- Guo J

journal: Frontiers in oncology

journalAbbr: Front Oncol

year: 2026

pubdate: 2026

volume: 16

issue:

pages: 1892580

abstract: Malignant melanoma, an aggressive neoplasm with high metastatic potential, rarely metastasizes to the breast, with bilateral involvement being exceptionally uncommon. This case report details a 36-year-old postpartum woman who presented with bilateral, symmetrical, rock-hard breast swelling and erythema, initially misdiagnosed as severe lactational mastitis. Comprehensive evaluation ultimately revealed diffusely metastatic malignant melanoma. The primary site was identified as a previously excised cutaneous lesion on the chest wall, originally reported as a benign compound nevus. The patient exhibited a rapidly progressive and refractory disease course, failing to respond to sequential targeted therapy, immunotherapy, and chemotherapy, and succumbed to the disease within seven months of diagnosis. This case underscores a profound diagnostic pitfall, highlighting the potential for metastatic melanoma to mimic benign inflammatory breast conditions, particularly in the physiologically altered postpartum period. It emphasizes the critical need for a high index of suspicion for metastatic disease in atypical breast presentations and reinforces the imperative of meticulous history-taking, including a thorough review of all prior skin pathology, in patients with malignancy of unknown origin. The aggressive clinical course further illustrates the formidable therapeutic challenges in such advanced presentations of metastatic melanoma.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13381241/

Record 82

pmcid: PMC13246418

pmid: 42272651

doi: 10.3389/fonc.2026.1778276

title: Metastatic malignant phyllodes tumor of the breast with rapid progression and subsequent response to immunotherapy: a case report.

authors

- Cao L
- Hu J
- Mao X
- Xu Y
- Ma W
- She X
- Zhang Y
- Yang P
- Zheng X

journal: Frontiers in oncology

journalAbbr: Front Oncol

year: 2026

pubdate: 2026

volume: 16

issue:

pages: 1778276

abstract: BACKGROUND: Malignant phyllodes tumors (MPTs) of the breast are rare fibroepithelial neoplasms characterized by aggressive clinical behavior and limited evidence guiding systemic therapy in the metastatic setting. Immune checkpoint inhibitors (ICIs) have demonstrated activity in several sarcoma subtypes; however, their role in MPTs of the breast remains poorly defined. CASE PRESENTATION: A 53-year-old female patient underwent modified radical mastectomy for a rapidly enlarging left breast mass. Pathology revealed a 16.0 × 15.0 × 9.5 cm malignant phyllodes tumor with lymphovascular invasion (pT4N0). Despite the presence of an 8-mm pulmonary nodule at baseline, multiple bilateral lung metastases developed shortly after surgery. Following eight cycles of ifosfamide plus epirubicin chemotherapy, tumor burden was reduced by more than 90%. The patient subsequently received anlotinib as maintenance therapy; four months later, new hepatic and colorectal metastases emerged while the pulmonary lesions remained stable. Second-line sintilimab-based combination therapy achieved a marked partial response, with significant regression of the abdominal lesions, liquefactive necrosis of the hepatic metastases, and complete relief of abdominal pain. CONCLUSION: This case illustrates the diagnostic and therapeutic complexity of metastatic malignant phyllodes tumors. Although first-line chemotherapy produced marked pulmonary tumor regression, subsequent anlotinib maintenance therapy did not prevent disease progression. In this PD-L1-positive tumor (CPS 10), sintilimab-based combination therapy yielded a marked clinical and radiographic response, supporting further investigation of ICI-based therapy in selected patients with metastatic MPT.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13246418/

Record 83

pmcid: PMC13481097

pmid: 42517198

doi: 10.1093/asj/sjag105

title: Mid- to Long-term Outcomes Following Primary Augmentation With B-Lite Lightweight Breast Implants: A Prospective Multicenter Postmarketing Clinical Follow-up Study.

authors

- Reichenberger M
- Hedén P
- Germann G

journal: Aesthetic surgery journal

journalAbbr: Aesthet Surg J

year: 2026

pubdate: 2026 Aug 18

volume: 46

issue: 9

pages: NP88-NP96

abstract: BACKGROUND: Lightweight breast implants (B-Lite) integrate air-filled microspheres into silicone gel to reduce implant weight by up to 30% while maintaining volume and mechanical properties. Long-term prospective data in primary augmentation remain limited. OBJECTIVES: To evaluate clinical performance, safety, satisfaction, and quality of life (QoL) outcomes following primary augmentation with B-Lite implants. METHODS: This multicenter, prospective, single-arm manufacturer-sponsored postmarketing clinical follow-up (PMCF) study enrolled genetically female patients (18-60 years) undergoing bilateral primary breast augmentation. Implants (round or anatomical; POLYsmoooth, POLYtxt, MESMO, or Microthane; 200-755 cc) were placed through inframammary incisions using a standardized technique. The primary endpoint was ≥1 bra-cup increase at 12 months; secondary endpoints included QoL (Body Esteem, Rosenberg, shortened Tennessee, RAND SF-36), satisfaction, and adverse events (AEs) such as capsular contracture (Baker III-IV), rupture, breast implant-associated anaplastic large cell lymphoma (BIA-ALCL), and reoperation. RESULTS: In all, 101 patients (mean age 32.7 ± 8.0 years; BMI 20.9 ± 1.8) underwent augmentation. At 12 months, ≥1-cup increase occurred in 92% (95% CI, 84%-97%). No Baker III/IV contracture, rupture, or BIA-ALCL occurred. The most frequent AE was transient sensory change (8.9%); others were infrequent and mainly procedure related. Kaplan-Meier reoperation risk was 6.9% at 1 year and 9.9% at 5 years. Patient satisfaction averaged 98.6% with 0% regret; surgeons were satisfied or very satisfied at 97% to 100%. QoL showed durable gains in Body Esteem-sexual attractiveness and Rosenberg; SF-36 composites remained stable. CONCLUSIONS: Lightweight B-Lite implants showed high effectiveness, very high satisfaction, and a favorable safety profile through 3 to 5 years. LEVEL OF EVIDENCE: 2: For image description, please refer to the figure legend and surrounding text.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13481097/

Record 84

pmcid: PMC13130240

pmid: 42056760

doi: 10.1080/0886022X.2026.2656561

title: Mineralocorticoid receptor antagonists in dialysis patients after ACHIEVE and ALCHEMIST: updated systematic review and meta-analysis of 14 randomized trials.

authors

- Zhu Y
- Li J
- Huang Z
- Qiu F

journal: Renal failure

journalAbbr: Ren Fail

year: 2026

pubdate: 2026 Dec

volume: 48

issue: 1

pages: 2656561

abstract: End-stage kidney disease (ESKD) patients receiving dialysis bear a heavy burden of cardiovascular disease (CVD), the leading cause of mortality. Mineralocorticoid receptor antagonists (MRAs) have cardiovascular protective effects in non-dialysis patients, but their efficacy and safety in dialysis-dependent individuals remain controversial. This study aimed to clarify their clinical value via an updated systematic review and meta-analysis. Following preferred reporting items for systematic reviews and meta-analyses (PRISMA) guidelines, we systematically searched PubMed, EMBASE, and The Cochrane Library up to 25 August 2025. Eligible studies were parallel-design randomized controlled trials (RCTs) enrolling adults (≥18 years) on maintenance dialysis, comparing MRAs with placebo or no intervention. Two reviewers independently screened studies, extracted data, and assessed quality using the Cochrane Risk of Bias Tool 2.0. Meta-analysis was performed with Cochrane Review Manager 5.4. The results showed that a total of 14 RCTs involving 4,525 patients were included (13 used spironolactone, one used eplerenone; follow-up: 3-40.8 months). MRAs significantly reduced nonfatal CVD events (RR 0.68, p = 0.04) but had no significant effect on cardiovascular mortality (RR 0.75, p = 0.14) or all-cause mortality (RR 0.76, p = 0.05). They significantly elevated risks of severe hyperkalemia (RR 1.35, p = 0.009) and gynecomastia and breast pain (RR 4.23, p < 0.001). In conclusion, MRAs can reduce the incidence of nonfatal cardiovascular events in dialysis patients; however, clinicians also need to be mindful of the risks of severe hyperkalemia as well as gynecomastia and breast pain in men when prescribing them.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13130240/

Record 85

pmcid: PMC13110349

pmid: 42046775

doi: 10.1155/ijfo/3848349

title: Modern Advances in Velvet Antler Research: From Identification Methods to Pharmacological Mechanisms.

authors

- Zhang M
- Liu X
- Jiang N

journal: International journal of food science

journalAbbr: Int J Food Sci

year: 2026

pubdate: 2026

volume: 2026

issue:

pages: 3848349

abstract: Velvet antler, a valued traditional medicine of animal origin, contains an abundance of bioactive compounds including amino acids, peptides, proteins, and nucleosides. It exhibits notable health-promoting properties such as wound repair, antioxidant activity, and metabolic regulation, making it a high-quality natural ingredient for functional foods and biopharmaceutical applications. Currently, velvet antler has gained widespread global use as a dietary supplement. Nevertheless, challenges remain in species authentication, mechanistic understanding of active constituents, and quality standardization, which impede the modern

development and efficient utilization of this resource. This review summarizes recent advances in identification techniques for velvet antler-encompassing traditional, chemical, and molecular methods-analyzes its chemical composition and influencing factors, and explores the potential mechanisms behind its various pharmacological effects. Furthermore, key issues concerning safety evaluation, quality control standardization, and industry regulation are critically discussed. The findings provide a solid theoretical basis for improving the quality consistency and application reliability of velvet antler products, with the aim of promoting the scientific development and global industrialization of velvet antler resources in food science and biomedicine.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13110349/

Record 86

pmcid: PMC13406403

pmid: 42512319

doi: 10.3390/cancers18142254

title: Multidisciplinary Management of Complicated Locally Advanced and Fungating Breast Cancer: A Narrative Review.

authors

- Obleagă CV
- Florescu MM
- Florescu LM
- Ahmet RAM
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- Mehedinţeanu AM
- Schenker M
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journal: Cancers

journalAbbr: Cancers (Basel)

year: 2026

pubdate: 2026 Jul 14

volume: 18

issue: 14

pages:

abstract: CONTEXT: Locally advanced fungating breast cancer complicated by hemorrhage or infection represents a complex clinical condition that often requires a rapid and coordinated multidisciplinary approach. Although international guidelines, such as those developed by ESMO and NCCN, provide well-defined principles for the management of locally advanced breast cancer, they offer limited recommendations regarding the management of complicated fungating forms. METHODS: We conducted a structured narrative review of the literature using the PubMed, Scopus, and Web of Science databases, including studies published between December 2019 and March 2026. Studies addressing locally advanced breast cancer and fungating forms complicated by hemorrhage or infection were included. RESULTS: Available data are heterogeneous and derive primarily from case reports, small case series and retrospective studies. Systemic therapy remains the cornerstone of oncologic control; however, the presence of acute complications often necessitates prioritization of local interventions depending on the patient's clinical stability. Hemorrhage may be managed through arterial embolization, hemostatic dressings or radiotherapy, whereas infection requires local wound care and targeted antibiotic therapy. These interventions aim to control acute symptoms and stabilize patients, thereby facilitating the subsequent integration of oncologic treatment. Emergency surgical intervention is reserved for selected cases, particularly when conservative measures fail to control bleeding or in the presence of persistent sepsis. CONCLUSIONS: The management of fungating breast cancer should be individualized and guided by the

clinical presentation within a multidisciplinary team setting rather than by a rigid therapeutic sequence. This paper proposes a pragmatic, symptom-oriented clinical framework integrating all currently available therapeutic modalities (systemic therapy, radiotherapy, interventional radiology, surgery, and supportive care) with the aim of supporting decision-making in complex clinical scenarios.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13406403/

Record 87

pmcid: PMC13449916

pmid: 42569372

doi: 10.1016/j.radcr.2026.05.085

title: Multimodality imaging features of primary mammary angiosarcoma: a case report with delayed osseous metastases.

authors

- Alsaleh B
- Alanzi A
- Alatefi D
- Alkabazi M
- Aldossary HH
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- Alsaeed M
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journal: Radiology case reports

journalAbbr: Radiol Case Rep

year: 2026

pubdate: 2026 Oct

volume: 21

issue: 10

pages: 5011-5021

abstract: Primary angiosarcoma of the breast is an uncommon and aggressive vascular malignancy that typically affects young women and poses diagnostic and therapeutic challenges. We report the case of a 26-year-old nulliparous woman who presented with progressive left breast swelling over approximately 18 months. Clinical examination revealed diffuse tender swelling without skin discoloration, nipple changes, or palpable lymphadenopathy. Ultrasonography demonstrated a complex cystic mass with thick septations and increased vascularity, while mammography showed global asymmetry of the left breast without suspicious calcifications. Breast MRI revealed a large 8.4 cm heterogeneously enhancing mass in the upper outer quadrant with possible focal skin involvement, but no chest wall invasion or nodal disease. Initial staging with 18F-FDG PET/CT showed no distant metastases. The patient underwent left nipple-sparing mastectomy, and histopathological examination confirmed intermediate-grade primary mammary angiosarcoma, FNCLCC grade 2. Subsequent re-excision achieved clear surgical margins. She received adjuvant ifosfamide-epirubicin chemotherapy and radiotherapy. Initial follow-up imaging showed no residual or recurrent disease. However, 14 months after completion of treatment, follow-up 18F-FDG PET/CT demonstrated new FDG-avid lytic lesions in the sacral and iliac bones, and Breast MRI revealed a small enhancing sternal lesion, consistent with delayed osseous metastatic disease. The patient was treated with palliative radiotherapy and systemic therapy. This case highlights the diagnostic complexity of primary breast angiosarcoma, its potential for delayed hematogenous osseous spread, and the value of multimodality imaging in diagnosis, staging, treatment planning, and surveillance.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13449916/

Record 88

pmcid: PMC13578126

pmid: 42742673

doi: 10.1007/s12550-026-00679-5

title: Mycotoxin contamination in food and feed: Occurrence, co-occurrence, combined impacts on human and animal health, climate change implications, risk assessment, and mitigation strategies.

authors

- Adelusi OA
- Okechukwu VO
- Odukoya JO
- Changwa R
- Aasa AO
- Oladeji OM
- Nwulu N
- Kondiah K
- Adebo OA
- Obadina AO
- El-Mesery HS
- De Saeger S
- Njobeh PB

journal: Mycotoxin research

journalAbbr: Mycotoxin Res

year: 2026

pubdate: 2026 Sep 15

volume: 42

issue: 4

pages:

abstract: Mycotoxins are toxic secondary metabolites produced by numerous fungi that infect crops during pre- and post-harvest stages. Aflatoxins, ochratoxins, fumonisins, zearalenone, and trichothecenes are of greatest concern due to their widespread occurrence and high toxicity. Consumption of mycotoxin-contaminated food and feed poses serious public health risks, as these compounds are linked to carcinogenic, neurotoxic, nephrotoxic, hepatotoxic, and immunotoxic effects. The frequent co-occurrence of multiple mycotoxins further amplifies health risks through additive or synergistic interactions and contributes to substantial economic losses. This review provides an updated overview of the global distribution and co-occurrence of major mycotoxins in agricultural products, their individual and combined toxicological effects on human and animal health, and the regulatory standards and maximum permitted levels for mycotoxins in food and feed. In addition, this review discusses the impact of climate change on mycotoxin contamination in food and feed, exposure, and associated health risks. It further outlines mycotoxin risk assessment procedures and highlights mitigation strategies for reducing contamination and limiting human and animal exposure risks. Ultimately, this review underscores the urgent need for integrated mycotoxin risk management strategies and strengthened global surveillance systems to safeguard food safety, protect human and animal health, and enhance food security.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13578126/

Record 89

pmcid: PMC12936644

pmid: 40862488

doi: 10.1002/ijgo.70451

title: Non-pharmacologic therapies for treating sexual dysfunction during pregnancy: A systematic review and meta-analysis.

authors

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journal: International journal of gynaecology and obstetrics: the official organ of the International Federation of Gynaecology and Obstetrics

journalAbbr: Int J Gynaecol Obstet

year: 2026

pubdate: 2026 Mar

volume: 172

issue: 3

pages: 1317-1334

abstract: BACKGROUND: Sexual dysfunction during pregnancy is a common problem, and can occur as the result of various physical, hormonal, and emotional changes that women experience during this period. Non-pharmacologic treatments are recommended due to the restrictions on medications during this period. OBJECTIVES: To summarize the evidence on non-pharmacologic interventions for treating sexual dysfunction during pregnancy. SEARCH STRATEGY: Searches were conducted in PubMed, Scopus, Web of Science, Embase, PsycINFO, PEDro, the Cochrane Central Register of Controlled Trials, and ClinicalTrials.gov. SELECTION CRITERIA: We included randomized clinical trials comparing interventions for treating sexual dysfunction during pregnancy. DATA COLLECTION AND ANALYSIS: From these search results, studies were selected, and data was extracted by two authors independently. The risk of bias was assessed using the Cochrane Risk of Bias tool (RoB 2.0). RevMan 5.4. was used for data synthesis. The Grading of Recommendations Assessment Development (GRADE) and Evaluation method was used to evaluate the strength of the evidence. MAIN RESULTS: We retrieved a total of 9017 articles. Twenty-four studies met the eligibility criteria and were included in the systematic review, and nine studies were included in the meta-analysis. The total number of participants was 1557, with a mean age ranging from 19.3 to 30.7 years and a gestation ranging from 8.9 to 28.3 weeks. Patients undergoing cognitive behavioral therapy (CBT) had a mean increase of 6.82 in their Female Sexual Function Index (FSFI) total score (range 1.63-12.01, P = 0.010, I 2 = 3%). For the Permission, Limited Information, Specific Suggestions, and Intensive Therapy (PLISSIT) model, the mean total FSFI score was 6.07 points higher (range 3.80-8.35, P = 0.00001, I 2 = 80%), and for the sex education intervention, the mean total score was 5.82 points higher (range 4.19-7.46, P = 0.00001, I 2 = 81%) than the control group. CONCLUSION: CBT, the PLISSIT model, and sexual education for pregnant women can improve sexual function during the gestational period.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12936644/

Record 90

pmcid: PMC13626010

pmid: 42820094

doi: 10.2147/CCID.S630692

title: Oral and Topical Spironolactone in Adult Female Acne: A Narrative Review Comparing Systemic and Local Approaches.

authors

- Łyko M
- Złotowska A
- Jankowska-Konsur A

journal: Clinical, cosmetic and investigational dermatology

journalAbbr: Clin Cosmet Investig Dermatol

year: 2026

pubdate: 2026

volume: 19

issue:

pages: 630692

abstract: BACKGROUND: Adult female acne (AFA) affects 12-45% of women over 25 years of age and is frequently resistant to conventional therapies. The antiandrogenic properties of spironolactone have positioned it as a promising non-antibiotic alternative. While oral spironolactone has been increasingly studied, its topical formulation remains comparatively underexplored, and recent evidence syntheses have generally pooled both routes of administration rather than comparing them within a framework specific to AFA. OBJECTIVE: This narrative review synthesizes and critically compares the available evidence on the efficacy, safety, and practical considerations of oral versus topical spironolactone in AFA. METHODS: A literature search was conducted in PubMed and Google Scholar on 1 September 2025, using search terms combining adult female acne with spironolactone. Original clinical studies-randomized controlled trials, observational studies, and case series-were included. Preclinical studies, non-English-language articles, and non-original data were excluded. RESULTS: Oral spironolactone (50-200 mg/day) consistently demonstrates significant reductions in inflammatory and non-inflammatory lesion counts and improves acne-specific quality of life. Head-to-head trials indicate comparable or superior efficacy relative to doxycycline, with longer treatment persistence than oral antibiotics. Topical spironolactone (2-5%) reduces comedones, papules, and pustules with minimal systemic absorption, suggesting a potential option for patients who are not candidates for systemic therapy. However, the evidence base for topical formulations remains limited by small sample sizes and heterogeneous study designs, and is derived largely from small mixed-sex or non-AFA cohorts and from pharmacokinetic studies in healthy volunteers; its applicability to AFA is therefore currently indirect and extrapolated. CONCLUSION: Oral spironolactone is supported by robust evidence as an effective non-antibiotic option for AFA and is suitable for moderate-to-severe disease, whereas topical spironolactone appears promising but remains supported by limited and partly indirect evidence, derived largely from small and heterogeneous studies, and may be appropriate for mild-to-moderate acne or when systemic therapy is contraindicated. Because no head-to-head trial has compared the two routes, the comparison presented here is necessarily indirect. Future research should prioritize head-to-head comparisons of both routes and standardization of topical formulations.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13626010/

Record 91

pmcid: PMC13225575

pmid: 42232883

doi: 10.1097/GOX.0000000000007795

title: Outcomes and Complications of Autologous Fat Transfer for Total Breast Reconstruction and Augmentation: Systematic Review and Meta-analysis of Randomized Controlled Trials.

authors

- Meijer GS
- van den Eijkel RCM
- Smit JM
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- Young-Afat DA
- Negenborn VL

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2026

pubdate: 2026 Jun

volume: 14

issue: 6

pages: e7795

abstract: BACKGROUND: Autologous fat transfer (AFT) for total breast reconstruction and augmentation is increasingly applied due to its advantages over implants and autologous flaps, including minimal invasiveness, minimal donor-site morbidity, and quick recovery. However, AFT is not without complications when applied for full breast reconstruction, underscoring the need to evaluate its safety and effectiveness. This systematic review and meta-analysis assesses the oncological and surgical safety of AFT as a stand-alone procedure for total breast reconstruction and augmentation, including the effectiveness of different AFT techniques and patient-reported satisfaction. METHODS: A MEDLINE, Embase, and Web of Science search (from inception to December 2024) focusing solely on randomized controlled trials evaluating AFT outcomes for total breast reconstruction or augmentation was performed. Animal studies, partial reconstruction/augmentation, and combined procedures were excluded. Data were extracted and analyzed using random-effects meta-analyses, following PRISMA (Preferred Reporting Items for Systematic Reviews and Meta-Analysis) guidelines. Risk of bias was assessed using the Cochrane tool. RESULTS: From 2323 identified articles, 9 randomized controlled trials were included. AFT showed fewer surgical complications compared with implant-based reconstruction. Oncological safety was similar between AFT and implant-based reconstruction. AFT-related complications (eg, oil cysts, fat necrosis) occurred at similar rates across different AFT techniques, but using fat sedimentation and retropectoral grafting were associated with fewer complications. Meta-analysis showed that all enriched AFT techniques-including stromal vascular fraction and botulinum toxin-improved fat retention rates. CONCLUSIONS: AFT is an effective and technically and oncologically safe option for total breast reconstruction and total breast augmentation. However, limited follow-up duration prevents definitive conclusions on long-term oncological outcomes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13225575/

Record 92

pmcid: PMC13040969

pmid: 41742298

doi: 10.1186/s13104-026-07735-4

title: Patient-reported satisfaction after ultrasound-assisted liposuction with infra-areolar excision for gynecomastia: a retrospective cross-sectional from Palestine.

authors

- Aljamal M
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journal: BMC research notes

journalAbbr: BMC Res Notes

year: 2026

pubdate: 2026 Feb 25

volume: 19

issue: 1

pages:

abstract: BACKGROUND: Gynecomastia can have significant psychosocial impacts. Ultrasound-assisted liposuction (UAL) combined with glandular excision has emerged as a preferred technique for contour correction with minimal scarring. This study evaluates surgical outcomes and patient satisfaction following UAL with glandular excision. METHODOLOGY: This was a retrospective cross-sectional study of male patients who underwent bilateral gynecomastia correction using ultrasound-assisted liposuction with infra-areolar excision between September 2022 and June 2024 in Palestine. Clinical and perioperative variables were extracted from medical records, and outcomes were supplemented with patient-reported satisfaction using the validated Arabic BODY-Q chest and nipple modules collected via an online follow-up questionnaire. Nonparametric group comparisons, Spearman correlation, and multivariable linear regression were used to identify predictors of satisfaction (SPSS v21). RESULTS: A total of 133 patients were included (median age 28 years; median BMI 26 kg/m^2). Most cases were physiological and primarily Simon grade IIa/IIb. The median aspirated fat volume was 350 Ml. Postoperative complications were generally infrequent; the most common were asymmetry, redundant skin, hematoma, and contour irregularity, and 12% of patients underwent revision surgery. Overall satisfaction was high in both BODY-Q domains. Chest satisfaction decreased with increasing gynecomastia grade and was independently lower in patients with contour irregularity, undesirable scars, redundant skin, and recurrence. Nipple satisfaction was independently lower with contour irregularity, undesirable scars, redundant skin, and nipple-areola complex adherence. Chest and nipple satisfaction scores were positively correlated. CONCLUSION: Ultrasound-assisted liposuction with infra-areolar excision achieved high patient-reported satisfaction with a low-to-moderate complication profile in this Palestinian cohort. Satisfaction was driven primarily by postoperative aesthetic factors-particularly contour quality, scarring, redundant skin, and recurrence, highlighting key targets for surgical optimization and patient counselling.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13040969/

Record 93

pmcid: PMC13325677

pmid: 41340520

doi: 10.4274/jcrpe.galenos.2025.2025-10-3

title: Peer Victimization and Psychological Outcomes in Adolescents with Pubertal Gynecomastia: A Case-Control Study.

authors

- Çalışkan Y
- Kocabey Sütçü Z
- Aytaç Kaplan EH

journal: Journal of clinical research in pediatric endocrinology

journalAbbr: J Clin Res Pediatr Endocrinol

year: 2026

pubdate: 2026 Jul 1

volume: 18

issue: 2

pages: 296-304

abstract: OBJECTIVE: Pubertal gynecomastia is associated with psychosocial consequences including anxiety, depression, and body image disturbances. However systematic examination of bullying experiences and their psychological correlates in adolescents with gynecomastia remains limited. The aim of this study was to investigate peer victimization prevalence and its relationship with psychological outcomes in this vulnerable population. METHODS: This case-control study included male adolescents aged 10-17 years, comprising half with gynecomastia and half healthy controls. Gynecomastia diagnosis and severity were assessed using clinical examination and Rohrich classification. Participants completed validated Turkish versions of the Olweus Bully/Victim Questionnaire, Rosenberg Self-Esteem Scale, and Revised Child Anxiety and Depression Scale. RESULTS: A total of 155 adolescents were included, with 78 (50.3%) having gynecomastia and the remaining 77 being healthy. Adolescents with gynecomastia demonstrated significantly higher peer victimization rates compared to controls (34.6% versus 16.9%, p=0.012), with markedly increased victim-perpetrator status (12.8% versus 1.3%, p=0.005). Gynecomastia diagnosis increased victimization risk 2.63-fold (95% confidence interval: 1.076-6.436, p=0.034). Victimized participants exhibited elevated anxiety and depression scores across multiple symptom domains (p<0.05). Behavioral modifications were prevalent, including altered clothing preferences (58.9%), changing room avoidance (44.8%), and swimming avoidance (41.0%). CONCLUSION: Adolescents with gynecomastia experienced substantially elevated peer victimization with consequential psychological impact in this cohort. These findings suggest the importance of routine bullying assessment during clinical evaluation and implementation of comprehensive psychosocial screening protocols with early intervention strategies.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13325677/

Record 94

pmcid: PMC13566113

pmid: 42730087

doi: 10.7759/cureus.114397

title: Persistent Secondary Hypogonadism Following Chronic Opioid and Anabolic Steroid Exposure: A Case Report of Long-Term Testosterone Replacement Therapy and Its Complications.

authors

- Hollis E
- Neamtu R
- Sandoval A
- Bruce K

journal: Cureus

journalAbbr: Cureus

year: 2026

pubdate: 2026 Aug

volume: 18

issue: 8

pages: e114397

abstract: Secondary hypogonadism is a clinically significant consequence of hypothalamic-pituitary-gonadal (HPG) axis suppression and may result from multiple exposures, including chronic opioid and anabolic steroid use. Long-term treatment strategies for persistent secondary hypogonadism, particularly testosterone replacement therapy (TRT) and its complications, remain incompletely characterized. This case describes a male patient with a history of both opioid and anabolic steroid exposure who developed persistent acquired secondary hypogonadism, with potential contributions from both opioid-induced androgen deficiency (OPIAD)

and anabolic steroid-induced hypogonadism (ASIH). The patient subsequently underwent approximately 13 years of TRT, with sustained symptomatic benefit but clinically significant complications including erythrocytosis and late-onset gynecomastia. At the time gynecomastia was documented, serum estradiol testing and breast imaging were not available in the records reviewed, and the diagnosis was based primarily on patient-reported symptoms. This case highlights the challenges of determining the etiology of persistent HPG axis suppression following multiple suppressive exposures and emphasizes the importance of longitudinal monitoring for complications associated with prolonged TRT.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13566113/

Record 95

pmcid: PMC13068176

pmid: 40059384

doi: 10.1111/jocn.17718

title: Perspectives and Requirements of Patients with Prostate Cancer on Mobile Health Interventions During Androgen Deprivation Therapy: A Descriptive Qualitative Study.

authors

- Yin H
- Pan C
- Gu J
- Yang Y

journal: Journal of clinical nursing

journalAbbr: J Clin Nurs

year: 2026

pubdate: 2026 May

volume: 35

issue: 5

pages: 2400-2415

abstract: AIM: To explore the perceptions of patients with prostate cancer undergoing androgen deprivation therapy towards mobile health interventions. DESIGN: The study employed a qualitative descriptive design. METHODS: Seventeen participants were recruited from tertiary care hospitals from November 2022 to June 2023. The collected data were subsequently analysed using a content analysis approach. RESULTS: The main themes were facilitators, barriers, information needs, emotional and social needs, and self-management needs. Facilitators for using mobile health included support from healthcare professionals and family, competitive design features and user-friendly interfaces. Barriers included negative past experiences with technology and a satisfactory current state of health. Informational support included those patients who expressed a desire for timely, accurate information integrated with traditional Chinese medical principles. Regarding emotional support, patients emphasised their independence, preferring not to burden family and friends, showing disinterest in mindfulness and relaxation therapies, and generally disliking online interactions with fellow patients. For self-management, medication reminders and rehabilitation guidance were seen as vital tools to enhance supportive care. CONCLUSION: The study highlights the importance of customising mobile healthcare applications to meet unique needs among patients with prostate cancer and enhance their engagement and satisfaction. Understanding their specific preferences enables the development of more effective mobile healthcare applications. REPORTING METHOD: The COREQ checklist.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13068176/

Record 96

pmcid: PMC13337816

pmid: 42415794

doi: 10.3389/fmed.2026.1797331

title: Pharmacovigilance profiles of three generations of mineralocorticoid receptor antagonists and network toxicology analysis.

authors

- Lin HH
- Cui WH
- Xu F
- He JY
- Li LC
- Xu JJ
- Jiang X
- Mao KL

journal: Frontiers in medicine

journalAbbr: Front Med (Lausanne)

year: 2026

pubdate: 2026

volume: 13

issue:

pages: 1797331

abstract: INTRODUCTION: Mineralocorticoid receptor antagonists, spironolactone, eplerenone, and finerenone, are cornerstone therapies in cardiovascular and renal diseases. While their efficacy is established, a systematic, head-to-head comparison of their post-marketing safety profiles based on large-scale real-world data is lacking. METHODS: This study comprehensively compared the post-marketing safety profiles of spironolactone, eplerenone, and finerenone using 11,556 primary suspect reports from the FAERS database (Q1 2004-Q3 2025). RESULTS: Disproportionality analyses revealed distinct adverse event (AE) signatures. Spironolactone exhibited the broadest safety profile, with disproportionate reporting indicating congenital malformations, acute pulmonary edema, serious cutaneous reactions, and antiandrogenic effects including rare signals such as endometriosis in males. Eplerenone showed a more focused yet severe profile, with signals for serious cardiac events and renal/hepatic failure. Finerenone demonstrated the most targeted profile, with signals predominantly confined to renal and metabolic domains. Hyperkalemia was a consistent moderate-priority signal across all three drugs, and several high-signal, unlabeled AEs were identified. Network toxicology analysis further supported these findings by linking shared and drug-specific molecular targets to key pathways (e.g., aldosterone-regulated sodium reabsorption, MAPK signaling). DISCUSSION: These hypothesis-generating findings underscore the need for personalized safety monitoring, potential label updates, and independent validation in other pharmacovigilance databases before clinical or regulatory conclusions can be drawn.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13337816/

Record 97

pmcid: PMC12909171

pmid: 41710411

doi: 10.3389/fendo.2026.1734644

title: Potential association between endocrine disrupting chemicals (EDCs) and gynecomastia: a systematic review based on partial experimental evidenceendocrine-disrupting chemicals.

authors

- Wang H
- Wang M
- Fan C
- Sun Y
- Feng J
- He M
- Li N
- Shang F
- Liu B

journal: Frontiers in endocrinology

journalAbbr: Front Endocrinol (Lausanne)

year: 2026

pubdate: 2026

volume: 17

issue:

pages: 1734644

abstract: Gynecomastia (GYN), the most prevalent benign breast condition in men, is primarily driven by an estrogen-androgen imbalance, which induces glandular proliferation and adipose hypertrophy. This imbalance leads to the proliferation of mammary gland tissue and hypertrophy of adipose tissue. Endocrine-disrupting chemicals (EDCs), as a class of exogenous substances widely distributed in the environment, can disrupt hormonal homeostasis by mimicking estrogen, antagonizing androgens, or interfering with hormone metabolism. Consequently, they represent a significant environmental risk factor for inducing male breast development. Some evidence also suggests that gynecomastia may represent a chronic, non-infectious inflammatory response to estrogenic stimulation, which could further alter the tissue's hormonal sensitivity. This paper systematically reviews typical EDCs associated with male breast development, including bisphenols, phthalates, and polycyclic aromatic hydrocarbons. It examines their exposure pathways and mechanisms of action, analyzes the clinical characteristics, public health implications, and current prevention and control status of gynecomastia. The review highlights existing issues in current research, such as unclear mechanisms and the complexity of mixed exposure effects. It proposes that future efforts should focus on strengthening research into the molecular mechanisms linking EDCs to male breast development, improving population exposure monitoring systems, and refining prevention and control strategies. This will provide a theoretical basis for the scientific prevention and clinical management of the condition. Furthermore, the paper emphasizes the critical importance of strictly controlling the environmental release of EDCs to protect public health.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12909171/

Record 98

pmcid: PMC13153758

pmid:

doi: 10.5114/pedm.2026.159282

title: Precocious puberty and preserved growth after craniopharyngioma treatment: case report and literature review.

authors

- Soloch M
- Robakowska K
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- Hylińska M

- Janeczek K
- Dyrka K
- Katarzyna Jończyk- P
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journal: Pediatric Endocrinology, Diabetes, and Metabolism

journalAbbr: Pediatr Endocrinol Diabetes Metab

year: 2026

pubdate: 2026 Mar

volume: 32

issue: 1

pages: 55-64

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13153758/

Record 99

pmcid: PMC13530398

pmid: 42682985

doi: 10.1155/crra/8222696

title: Primary Breast Involvement in Erdheim-Chester Disease: A Rare Presentation.

authors

- Morkos K
- Williams B
- Simpson I
- Ooi C
- Sawyer B

journal: Case reports in radiology

journalAbbr: Case Rep Radiol

year: 2026

pubdate: 2026

volume: 2026

issue:

pages: 8222696

abstract: Erdheim-Chester disease (ECD) is a rare multisystem histiocytic neoplasm. Breast manifestations of this condition are exceedingly rare with fewer than 20 cases reported in the literature. We present a case report of a 40-year-old female patient with a 5-year history of breast enlargement and pigmentation who was diagnosed with ECD involving the bones, retroperitoneum and breast, highlighting key imaging and pathologic features that led to the diagnosis. ECD should be considered in the differential diagnosis of infiltrative breast disease. Considering this differential diagnosis allows appropriate further assessment and management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13530398/

Record 100

pmcid: PMC13459501

pmid: 42582869

doi: 10.1093/omcr/omag149

title: Primary Burkitt lymphoma of the breast in a young north African woman: diagnostic challenge and benefit of early intensive chemotherapy.

authors

- Ibtihal A
- Bouanani N

journal: Oxford medical case reports

journalAbbr: Oxf Med Case Reports

year: 2026

pubdate: 2026 Aug

volume: 2026

issue: 8

pages: omag149

abstract: Burkitt lymphoma (BL) is a highly aggressive B-cell malignancy that rarely presents primarily in the breast. Its rapid progression and inflammatory features can mimic breast cancer or mastitis, leading to a delay in diagnosis. A 26-year-old nulliparous North African woman presented with a painful, rapidly growing breast mass that had appeared within the last four weeks. Clinical examination revealed an 8 cm mass accompanied by orange-peel skin. Imaging suggested a likely benign lesion (BI-RADS 3). However, histology showed a classic 'starry sky' pattern, a Ki-67 rate of 100% and positivity for CD20/CD10/BCL6 with negativity for BCL2/CD5/EBER. PET-CT revealed a solitary hypermetabolic lesion (SUV 32) without systemic involvement (stage IE, BL-IPI 1). Treatment with R-COPADM was initiated within 48 hours, resulting in complete remission. This case highlights that an early biopsy is essential in the presence of any atypical inflammatory breast mass, even when imaging appears reassuring.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13459501/

Record 101

pmcid: PMC13602233

pmid: 42787887

doi: 10.1016/j.radcr.2026.08.084

title: Primary lymphoma of the breast: A case report.

authors

- Sarah L
- Fatima Ez-Zahrae EM
- Laamrani FZ
- Omor Y
- Latib R
- Amalik S

journal: Radiology case reports

journalAbbr: Radiol Case Rep

year: 2026

pubdate: 2026 Dec

volume: 21

issue: 12

pages: 6202-6205

abstract: Primary breast lymphoma is a rare extranodal manifestation of non-Hodgkin lymphoma, accounting for less than 0.5% of all breast malignancies and approximately 1-2% of extranodal lymphomas. Although diffuse large B-cell lymphoma is the most frequently reported subtype, other indolent B-cell lymphomas can also occur. We present the case of a 70-year-old woman referred for evaluation of a left breast mass initially suspected to represent granulomatous mastitis on ultrasound. Histopathological examination with immunohistochemistry established the diagnosis of primary small B-cell follicular lymphoma grade 1-2 of the breast. This case highlights the diagnostic challenge posed by this uncommon entity and emphasizes the essential role of tissue biopsy and immunophenotyping.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13602233/

Record 102

pmcid: PMC13279689

pmid: 42328653

doi: 10.3389/fphar.2026.1820644

title: Pueraria mirifica: a critical review of the ethnobotany, phytochemistry, pharmacology and challenges in standardization and safety.

authors

- Zeng H
- Wen J
- Chen M
- Qiu Z
- Huang S
- Hao E
- Du Z
- Yao C
- Hou X
- Deng J

journal: Frontiers in pharmacology

journalAbbr: Front Pharmacol

year: 2026

pubdate: 2026

volume: 17

issue:

pages: 1820644

abstract: BACKGROUND: Pueraria candollei var. mirifica (Airy Shaw and Suvat.) Niyomdham (Pueraria mirifica), locally known as "White Kwao Krua" or "white kudzu", is a renowned medicinal botanical drug indigenous to Southeast Asia, particularly Thailand and Myanmar. It has long been utilized in ethnomedicine for estrogen-related conditions and age-associated functional decline. This review critically evaluates the nexus between its botanical characteristics, traditional ethnopharmacological uses, and contemporary evidence regarding its therapeutic potential. MATERIALS AND METHODS: A structured literature search was conducted across multiple international and Chinese databases up to 2025, with screening based on predefined inclusion and

exclusion criteria focusing on botanical identity, chemical composition, pharmacological activity, toxicological evidence, and clinical findings. Taxonomic validity and geographic distribution were verified using Plants of the World Online (POWO) and the Global Biodiversity Information Facility (GBIF). The names and chemical formulas of secondary metabolites were verified using the PubChem database, and the structures were drawn using ChemDraw 22.2. RESULTS: Phytochemical studies on the rhizomes of P. mirifica showed that it contains 106 kinds of secondary metabolites, including benzopyrans, isoflavones and their glycosides, coumarins, steroids, fatty acids and their derivatives, and other metabolites. Pharmacological evidence demonstrates that P. mirifica exhibits potent estrogenic, anti-osteoporotic, and antioxidant activities, alongside neuroprotective and immunomodulatory effects. Clinical data suggest that its high concentration of potent phytoestrogens effectively mitigates vasomotor symptoms and urogenital syndromes in menopausal women, while positively influencing lipid metabolism and bone mineral density. CONCLUSION: P. mirifica represents a high-potency phytoestrogenic candidate for menopause management. Nevertheless, the translation from traditional use to clinical standardization is hindered by a lack of rigorous pharmacokinetic data and inconsistent quality control of metabolites. Future research should move beyond descriptive summaries toward mechanistic studies that define long-term safety margins and standardized dose-response relationships in order to ensure therapeutic reliability.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13279689/

Record 103

pmcid: PMC13111065

pmid: 42052486

doi: 10.3389/fonc.2026.1810319

title: Rare overlap of subtype and phenotype: HER2-positive pleomorphic invasive lobular carcinoma presenting with an inflammatory breast cancer phenotype: a case report and targeted treatment strategy.

authors

- Xiong Z
- Zhou G
- Zhou N
- Wang L
- Liu G
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- Lyu G

journal: Frontiers in oncology

journalAbbr: Front Oncol

year: 2026

pubdate: 2026

volume: 16

issue:

pages: 1810319

abstract: Pleomorphic invasive lobular carcinoma (PLC) is a rare and aggressive subtype of invasive lobular carcinoma. When it presents with an inflammatory breast cancer (IBC) phenotype, due to its rarity, this can lead to substantial diagnostic and therapeutic challenges. Herein, we report the case of a 53-year-old woman presenting with a rapidly enlarging left breast mass, diffuse edema, erythema, and pain; these symptoms have affected the entire breast over the course of one month. Imaging revealed diffuse parenchymal abnormalities and nodal involvement, while biopsy indicated PLC, histologic grade III, with HER2 amplification and complete absence of hormone receptors. Clinical and radiological assessment demonstrated nodal involvement (supraclavicular, cervical, and bilateral axillary) and distant osseous metastases, consistent with stage cT4N3M1

disease. The patient received first-line docetaxel, trastuzumab, and pyrotinib (PyroHT) and achieved a partial response; however, progression was observed after ten cycles. Following multidisciplinary review, her treatment was changed to trastuzumab deruxtecan (T-DXd), resulting in significant symptom relief and radiologic regression of the breast lesions. This case highlights the importance of early biopsy in rapidly progressive breast disease and that treatment for rare histologic subtypes, such as PLC, should be guided by molecular profiling rather than histology alone. Sequential HER2-targeted therapy provided meaningful clinical benefit, though resistance ultimately developed. We present the case of a 53-year-old woman with HER2-positive advanced PLC presenting with an IBC phenotype, providing insight into the diagnostic process and treatment course in this rare clinical scenario.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13111065/

Record 104

pmcid: PMC13107182

pmid: 42039109

doi: 10.1210/jcemcr/luag064

title: Rare β-human chorionic gonadotrophin-secreting urothelial carcinoma initially presenting as gynecomastia.

authors

- Hoogstraet GM
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- Koontz PW

journal: JCEM case reports

journalAbbr: JCEM Case Rep

year: 2026

pubdate: 2026 May

volume: 4

issue: 5

pages: luag064

abstract: Gynecomastia is a relatively common condition arising from disproportionate concentrations of estrogen and androgen. Most cases of bilateral gynecomastia in men are idiopathic or drug-induced but can rarely arise secondary to malignancy. We present the case of a 57-year-old male who presented with new-onset gynecomastia and recurrent episodes of hematuria. Laboratory testing revealed elevated estradiol, free testosterone, and β-human chorionic gonadotropin (β-hCG) with suppressed luteinizing hormone (LH) and follicle stimulating hormone (FSH), concerning for ectopic β-hCG secretion. Mammography, breast and testicular ultrasonography, and chest computed tomography demonstrated no evidence of breast, testicular, or lung malignancy. Concurrent evaluation of hematuria with cystoscopy identified a 6-cm tumor on the posterior bladder wall. The patient underwent transurethral resection of the bladder tumor and was diagnosed with papillary high-grade T1 urothelial carcinoma. The β-hCG immunohistochemical staining of the tumor was positive, and the patient's β-hCG level was undetectable 1 month postoperatively. A review of the literature revealed there are only a few reports detailing urothelial carcinomas capable of secreting β-hCG. To the best of our knowledge, this is the first case of a β-hCG-secreting papillary T1 urothelial carcinoma initially presenting as gynecomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13107182/

Record 105

pmcid: PMC12817282

pmid: 41567526

doi: 10.1002/ccr3.71891

title: Rarity to Remission: Complete Response of Primary Breast Diffuse Large B-Cell Lymphoma With Chemoimmunotherapy.

authors

- Jha A
- Ghosh V
- Gupta S
- Oli S
- Rayamajhi A
- Gupta D
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- Khanal B
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- Khadka S

journal: Clinical case reports

journalAbbr: Clin Case Rep

year: 2026

pubdate: 2026 Jan

volume: 14

issue: 1

pages: e71891

abstract: Primary Non-Hodgkin's lymphoma of the breast is rare, accounting for < 0.5% of breast cancers and ~2% of extranodal lymphomas. It often presents as a painless lump, mimicking carcinoma and complicating diagnosis. Case PresentationWe report a 65-year-old post-menopausal, hypertensive, and diabetic woman with a gradually enlarging left breast mass. Imaging revealed a suspicious lesion. Core biopsy suggested poorly differentiated carcinoma, but immunohistochemistry confirmed diffuse large B-cell lymphoma (DLBCL), activated B-cell subtype, with high Ki-67 (~90%). PET/CT showed localized disease. She received six cycles of CHOP chemotherapy, with rituximab added from the second cycle. Treatment was well tolerated. Follow-up PET/CT and biopsy demonstrated complete metabolic and pathological remission. ConclusionPrimary breast DLBCL is rare and easily misdiagnosed as carcinoma. Immunohistochemistry is therefore crucial for correct diagnosis. R-CHOP chemoimmunotherapy is the cornerstone of treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12817282/

Record 106

pmcid: PMC13340718

pmid: 42416423

doi: 10.1097/RC9.0000000000000596

title: Reconstruction of a rare case of bilateral juvenile gigantomastia at a tertiary hospital: a case report.

authors

- Rugendabanga EK
- Murhula GB
- Pompermaier L

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2026

pubdate: 2026 Jun

volume: 138

issue: 7

pages: 2606-2611

abstract: INTRODUCTION: Juvenile gigantomastia is a rare benign breast pathology that predominantly affects adolescents and women younger than 18 years of age. It is characterized by excessive, diffuse enlargement of the breast tissue (more than 1.5-2.5 kg per breast or constituting over 3% of total body weight), involving both glandular and stromal components. The histological picture consists of a juvenile fibroadenoma (JFA), which represents a distinct subtype of fibroepithelial breast tumor. The condition can cause severe physical discomfort, pain, posture abnormalities, recurrent skin infections, difficulties with mobility and personal hygiene, low self-esteem, and social stigma. CASE PRESENTATION: A 15-year-old female presented with a 1-year history of progressive bilateral breast enlargement without nipple discharge, systemic symptoms, or trauma. On local examination, the breasts were grossly enlarged, soft, and non-tender on palpation, with the skin overstretched and visible ulcerations. Modified Goldilocks reduction mammoplasty was performed after excluding other pathologies through a breast ultrasound and hormonology report. Histology reported JFA. DISCUSSION: JFA is a rare form of benign breast tumor, which manifests during puberty and leads to excessive breast development (gigantomastia) and its related discomfort. It is hypothesized that the causes are due to hypersensitivity of breast tissue to normal circulating estrogens, and hormonal therapy has been reported as a possible treatment, but surgery is considered the definitive treatment. Counseling on follow-up plans should be done to detect complications and recurrence. CONCLUSION: JFA patients seek care for physical disability and psychological distress. Surgical intervention is the definitive treatment, and histology confirms the diagnosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13340718/

Record 107

pmcid: PMC13330619

pmid: 42405308

doi: 10.1016/j.jpra.2026.04.010

title: Refined reduction mammaplasty for severe breast hypertrophy: Technique and outcomes of a modified free nipple-areolar complex graft approach.

authors

- Pimanchev PV
- Reshetov IV
- Oganezova AA
- Isakova YI
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- Mortada H

journal: JPRAS open

journalAbbr: JPRAS Open

year: 2026

pubdate: 2026 Sep

volume: 51

issue:

pages: 153-160

abstract: BACKGROUND: Severe breast hypertrophy presents a significant surgical challenge, especially in high body mass index (BMI) patients, due to the risks of poor projection, wound complications, and nipple-areolar complex (NAC) necrosis. This study evaluates a refined technique combining free NAC grafting with dual fascial fixation and minimal suture stabilization to optimize outcomes in this high-risk population. METHODS: We retrospectively reviewed patients with severe breast hypertrophy who underwent reduction mammaplasty using the described technique between 2019 and 2023. Surgical indication for free NAC grafting was based on the presence of Grade 3 ptosis with required NAC elevation exceeding 8 cm and BMI ≥29 kg/m², reflecting the high-risk nature of the cohort for NAC ischemia with pedicled techniques. The technique involved Wise pattern resection, dual fixation of medial and lateral pedicles to the superficial fascia, subdermal inverting sutures, and four-point fixation of a full-thickness free NAC graft. Postoperative outcomes were assessed clinically and photographically, focusing on breast projection, NAC survival, scar quality, and patient satisfaction. RESULTS: All 42 patients (mean age 42.2 years, mean BMI 35.5 kg/m²) had no total NAC graft loss and maintained satisfactory breast projection and shape. The mean resection weight per breast was approximately 1135 g (range: 560-1710 g); the minimum value (560 g) represented the smaller breast of an asymmetric patient whose contralateral resection was 1470 g. Minor complications included focal areolar hypopigmentation in 2 patients and minor marginal necrosis in 4 cases (9.5%), all managed conservatively. Despite loss of lactation function and erogenous sensitivity, all patients reported high satisfaction with the aesthetic outcomes. CONCLUSION: Reduction mammaplasty using a refined free nipple-areolar complex grafting approach with dual fascial fixation and minimal suture stabilization represents a reliable and reproducible option for managing severe breast hypertrophy in high body mass index patients. When applied within a standardized operative framework and individualized patient selection, this technique allows for consistent breast projection, durable contour, and high patient satisfaction with a low complication profile.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13330619/

Record 108

pmcid: PMC13661090

pmid: 42871819

doi: 10.14740/jmc5386

title: Reversible Thyroid Nodule in Van Wyk-Grumbach Syndrome: An Unusual Case Report.

authors

- AlBishi LA

journal: Journal of medical cases

journalAbbr: J Med Cases

year: 2026

pubdate: 2026 Nov

volume: 17

issue: 11

pages: 628-636

abstract: Van Wyk-Grumbach syndrome (VWGS) is a rare complication of longstanding, untreated primary hypothyroidism in children. It is characterized by isosexual precocious puberty, delayed bone age, and multicystic ovarian enlargement despite the presence of severe hypothyroidism. Although thyroid enlargement is common, a discrete thyroid nodule has not previously been documented in this syndrome. This study presents a 6-year-old Saudi girl who exhibited a 2-day history of vaginal hemorrhage and a history of breast development for 6 months. The physical examination demonstrated a palpable right thyroid nodule. The laboratory evaluation revealed a severe primary hypothyroidism, high thyroid autoantibodies, elevated prolactin levels, and prepubertal levels of basal gonadotropins. A thyroid ultrasound demonstrated a well-defined, hypoechoic nodule. Further testing revealed that the child had a delayed bone age, multi-cystic ovaries, and pituitary hyperplasia. After levothyroxine treatment, the patient's thyroid hormone levels normalized. In addition, there was progressive shrinkage of the thyroid nodule, pituitary hypertrophy, and ovarian cysts, along with significant

improvement in linear growth. This case adds to the previously unreported association of a reversible thyroid nodule with VWGS and highlights the importance of early diagnosis and appropriate management of severe hypothyroidism to prevent unnecessary or invasive diagnostic tests.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13661090/

Record 109

pmcid: PMC13247007

pmid: 42034827

doi: 10.1007/s12672-026-05072-3

title: Right breast invasive carcinoma and contralateral left axillary abscess representing a rare dual pathology.

authors

- Addagarla S
- Ranjan A
- Sathyakumar K

journal: Discover oncology

journalAbbr: Discov Oncol

year: 2026

pubdate: 2026 Apr 25

volume: 17

issue: 1

pages:

abstract: A 53-year-old female patient complained of bilateral breast enlargement with a palpable mass in the left axilla. Contrast-enhanced MRI revealed a suspicious lesion in right breast along with an ill-defined collection in the left axilla. Targeted ultrasound revealed an irregular hypoechoic lesion in the right breast along with collections in the left axilla. FNAC of the lesion in right breast revealed invasive carcinoma, while FNAC of the left axillary swelling yielded purulent material suggesting abscess. Cartridge-based nucleic acid amplification testing (CBNAAT) detected the presence of Mycobacterium tuberculosis complex DNA without resistance to rifampicin. The patient was then given antitubercular treatment. The patient then underwent a wide local excision of the suspicious lesion in the right breast. The histopathological examination revealed Grade 3 invasive carcinoma of no special type (NST). This is a rare case of the coexistence of breast carcinoma along with contralateral tuberculous axillary lymphadenitis, which clinically mimicked metastatic axillary lymphadenopathy. The clinical, radiological and pathological correlation is essential to avoid any misdiagnosis of the tumor.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13247007/

Record 110

pmcid: PMC13617274

pmid: 42807185

doi: 10.3389/fped.2026.1891119

title: Risk factors and preventive strategies for precocious puberty: a narrative review.

authors

- Lin JQ
- Huang TT
- Zhang JC

- Tang XY
- Chen Y
- Wu XF
- Zhang X
- Lei EZ

journal: Frontiers in pediatrics

journalAbbr: Front Pediatr

year: 2026

pubdate: 2026

volume: 14

issue:

pages: 1891119

abstract: Precocious puberty (PP) is a common pediatric endocrine disorder with rising global incidence, significantly compromising children's quality of life and long-term health. Current therapeutic options remain limited in applicability, leaving many patients inadequately controlled. Prevention targeting known risk factors is substantially more cost-effective than post-diagnosis treatment; however, the available evidence on PP risk factors remains fragmented. This review systematically synthesizes the risk factors for PP across seven age stages-preconception period, fetal period, infancy, toddlerhood, preschool age, school age, and adolescence-and eight domains: genetic predisposition and family history, maternal conditions and prenatal factors, delivery mode and early feeding patterns, dietary behaviors and nutritional status, individual lifestyle habits, family/school/community environments, early clinical signs and indicators, and pre-existing and idiopathic medical conditions. Corresponding preventive strategies, including optimizing parental reproductive health, promoting balanced nutrition, physical activity, and psychosocial well-being, and fostering multi-sector collaboration, are briefly discussed. This review aims to raise awareness of PP risk factors and facilitate the implementation of preventive strategies.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13617274/

Record 111

pmcid: PMC13588306

pmid: 42761705

doi: 10.15537/1658-3175.8849

title: Scoping Review of Mental Health Outcomes of Cosmetic Procedures Among the Saudi Population.

authors

- Alzahrani H

journal: Saudi medical journal

journalAbbr: Saudi Med J

year: 2026

pubdate: 2026 Oct

volume: 47

issue: 10

pages: 1601-1612

abstract: OBJECTIVE: To comprehensively examine the mental health outcomes associated with cosmetic procedures among the Saudi population, with a particular focus on psychological well-being, self-esteem, anxiety, body image, and quality of life. METHODS: This scoping review was conducted in accordance with the

Joanna Briggs Institute (JBI) methodology and reported following the Preferred Reporting Items for Systematic reviews and Meta-Analyses extension for Scoping Reviews guidelines. A structured search was performed across international databases, including PubMed, PsycINFO, Scopus, Web of Science, and Google Scholar, in addition to national Arabic databases. Studies investigating the psychological outcomes of both surgical (rhinoplasty and body contouring) and non-surgical cosmetic procedures among Saudi populations were included. A total of 19 studies involving approximately 9,705 participants met the eligibility criteria and were analyzed. RESULTS: The findings demonstrated that cosmetic procedures were frequently associated with improvements in self-esteem, body image satisfaction, social functioning, and overall quality of life. Several studies reported enhanced psychological well-being and increased confidence following cosmetic interventions. However, some individuals, particularly those with pre-existing psychological vulnerabilities, unrealistic expectations, or low baseline self-esteem, continued to experience anxiety, dissatisfaction, and psychological distress after the procedures. Variations in outcomes were influenced by procedure type, demographic factors, and psychosocial characteristics. CONCLUSION: Cosmetic procedures may contribute to positive psychological and quality-of-life outcomes among many individuals in Saudi Arabia; however, adverse psychological effects may persist in vulnerable groups. These findings emphasize the importance of comprehensive preoperative psychological assessment, realistic expectation management, and multidisciplinary support to optimize outcomes and minimize potential mental health risk.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13588306/

Record 112

pmcid: PMC13426201

pmid: 42540544

doi: 10.1016/j.radcr.2026.06.136

title: Secondary breast involvement by peripheral T-cell lymphoma, not otherwise specified.

authors

- Patel P
- Lotfalla M
- Menke D
- Maimone S

journal: Radiology case reports

journalAbbr: Radiol Case Rep

year: 2026

pubdate: 2026 Oct

volume: 21

issue: 10

pages: 4742-4747

abstract: Peripheral T-cell lymphoma not otherwise specified (PTCL-NOS) is an aggressive nodal lymphoma that rarely manifests as breast involvement and may mimic mastitis or inflammatory breast cancer. We describe a 74-year-old woman with 1 month of progressive left breast pain, erythema, swelling, and fever, initially treated as refractory mastitis, who was later found to have secondary breast involvement by PTCL-NOS. Mammography revealed diffuse left breast skin/trabecular thickening, and ultrasound demonstrated a breast fluid collection with axillary lymphadenopathy. Cross-sectional imaging demonstrated extensive cervical, axillary, thoracic, and abdominal lymphadenopathy with secondary venous and lymphatic congestion of the breast. Axillary and inguinal lymph node biopsies confirmed the diagnosis of PTCL-NOS. The case highlights the diagnostic challenge of lymphoma-related breast manifestation, emphasizes the importance of early biopsy, and underscores the role of multidisciplinary collaboration among breast imaging, dermatopathology, and hematology-oncology.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13426201/

Record 113

pmcid: PMC13322902

pmid: 42396141

doi: 10.3389/fmed.2026.1787706

title: Sexual dysfunction associated with 5α-reductase inhibitors in the treatment of androgenetic alopecia: a systematic review.

authors

- Złotowska A
- Jastrząb-Miśkiewicz B
- Krajewski PK

journal: Frontiers in medicine

journalAbbr: Front Med (Lausanne)

year: 2026

pubdate: 2026

volume: 13

issue:

pages: 1787706

abstract: INTRODUCTION: 5α-reductase inhibitors (5-ARIs) are most commonly used to treat benign prostatic hyperplasia (BPH) and androgenetic alopecia (AGA). Preclinical and clinical studies suggest that in patients with AGA treated with 5-ARIs, a subset reports sexual adverse events, including decreased libido, erectile dysfunction, and ejaculatory disorders. This review explores the development of sexual dysfunction in patients with androgenetic alopecia due to the use of 5-ARIs, emphasizing their importance in the clinical diagnosis of health disorders coexisting with hair loss. METHODS: A systematic review was conducted by searching electronic databases, including MEDLINE, Scopus, Web of Science and Google Scholar, according to the PRISMA guidelines. The search was limited to articles published in English and up to December 2025. Key search terms included "5α-reductase inhibitors" or "5-ARIs" or "finasteride" or "dutasteride" AND "side effects" or "sexual side effects" or "sexual" or "sexual dysfunction" AND "androgenetic alopecia" or "male pattern hair loss" or "female pattern hair loss." Data synthesis included findings from 41 studies, comprising 33 primary-evidence studies in AGA/MPHL/FPHL populations and 8 supporting-evidence studies providing pharmacokinetic, mixed-indication, or comparative context. RESULTS: 5-ARIs are effective therapies for androgenetic alopecia and are generally well tolerated. Across placebo-controlled RCTs evaluating oral finasteride 1 mg in men with AGA, sexual adverse events were reported in 1.9-6.7% of treated patients compared with 0.9-3.9% in placebo groups, with most events mild and reversible upon discontinuation. Topical finasteride 0.25% was associated with lower rates of sexual adverse events (2.8%) compared with oral finasteride (4.8%). For dutasteride 0.5 mg, sexual adverse events ranged from 4.1 to 12.0% across RCTs, compared with 4.0-5.0% in placebo groups. No sexual adverse effects were consistently reported in women treated with 5-ARIs for AGA. In most reports, the effects were transient and reversible after discontinuation. CONCLUSION: Overall, clinicians should counsel patients that most sexual side effects reported in controlled AGA studies are infrequent, mild, and reversible, but individual susceptibility varies. Shared decision-making, careful monitoring of sexual function, and further high-quality long-term studies-using standardized definitions of sexual dysfunction and persistence-are needed to quantify risks better and identify vulnerable subgroups.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13322902/

Record 114

pmcid: PMC13002691

pmid: 41171556

doi: 10.1007/s40264-025-01626-6

title: Sexual Dysfunctions Associated with Proton Pump Inhibitors: Insights from VigiBase, the World Health Organization Pharmacovigilance Database.

authors

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- Scapini F
- L'Abbate L
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journal: Drug safety

journalAbbr: Drug Saf

year: 2026

pubdate: 2026 Apr

volume: 49

issue: 4

pages: 435-447

abstract: BACKGROUND: Over the years, several observational studies and case reports have been published hypothesizing a potential association between the use of proton pump inhibitors (PPIs) and sexual dysfunctions in both male and female patients. OBJECTIVES: We aimed to investigate the potential association between PPI use and sexual dysfunction onset using VigiBase, the World Health Organization international pharmacovigilance database. METHODS: All individual case safety reports of sexual dysfunctions containing PPIs (i.e. omeprazole, lansoprazole, rabeprazole, pantoprazole and esomeprazole) as suspected or interacting drugs until 4 September, 2024 were selected from VigiBase using the Standardised MedDRA Query "Sexual dysfunction". A descriptive analysis of the selected individual case safety reports was carried out. Potentially new safety signals were identified through a disproportionality analysis and calculated as a reporting odds ratio (ROR) along with a 95% confidence interval (CI), which were adjusted via Bonferroni correction for multiple testing. To account for age-related effects on sexual functions, a subgroup analysis was carried out by restricting the study population only to patients aged from 18 to 64 years. RESULTS: A total of 420,598 individual case safety reports concerning PPIs were collected in VigiBase during the study period. Of these, 841 containing at least one Preferred Term included in the "Sexual dysfunction" Standardised MedDRA Query reporting information on sex were retrieved. Overall, disproportionate reporting for omeprazole concerning erectile dysfunction (adjusted ROR, 1.76; 95% CI 1.54-2.01) was observed, while two statistically significant adjusted RORs for esomeprazole, i.e. genital discomfort (ROR, 3.66; 95% CI 1.34-10.04) and oestrogen deficiency (ROR, 3.80; 95% CI 1.03-13.99) in female patients were found. The subgroup analysis confirmed the statistically significant disproportionate reporting of erectile dysfunction for omeprazole (adjusted ROR: 1.80; 95% CI 1.49-2.16), and generated new potential safety signals including an omeprazole-induced libido decrease (adjusted ROR, 1.49; 95% CI 1.05-2.12) and esomeprazole-induced hypogonadism (adjusted ROR, 5.22; 95% CI 1.22-22.34) in male individuals, and omeprazole-induced genital discomfort (adjusted ROR, 3.55; 95% CI 1.13-11.09) in female individuals. CONCLUSIONS: Findings of this study suggest the presence of safety signals of PPI-induced sexual dysfunctions, such as erectile dysfunction, genital discomfort and oestrogen deficiency. However, further observational studies are required to validate and further characterise these potential safety signals.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13002691/

Record 115

pmcid: PMC12830078

pmid: 41584304

doi: 10.1016/j.bbrep.2026.102448

title: Single-cell transcriptome sequencing revealing the microenvironment of breast fibroepithelial tumor.

authors

- Shu T
- Wang J
- Yang F
- Yang D
- Ren S
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- Xie H
- Yang X
- Jiang C
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journal: Biochemistry and biophysics reports

journalAbbr: Biochem Biophys Rep

year: 2026

pubdate: 2026 Mar

volume: 45

issue:

pages: 102448

abstract: BACKGROUND: Giant juvenile fibroepithelial tumors of the breast are rare lesions that mainly affect adolescent girls. Their pathogenesis is poorly understood, and current management relies almost entirely on surgery, which may interfere with breast development. To clarify the biological basis of these tumors, we performed single-cell RNA sequencing. METHODS: We profiled 15,295 cells from three juvenile cases (ages 12-16) and 11,442 cells from one adult patient (age 42) using the BD Rhapsody™ platform. This dataset provided a detailed cellular atlas of fibroepithelial lesions and enabled direct comparison of tumor microenvironments between juvenile and adult cases. RESULTS: Distinct molecular signatures in epithelial cells and fibroblasts: Juvenile FT epithelial cells exhibited significant enrichment of proliferation-associated pathways (e.g., PI3K-Akt signaling, $p = 3.2 \times 10^{-5}$; GnRH signaling, $p = 1.8 \times 10^{-4}$), whereas adult lesions were dominated by estrogen receptor pathways ($p = 2.6 \times 10^{-6}$). Juvenile-specific antigen-presenting fibroblasts : Comprising 18.7 % of stromal cells (vs. <1 % in adults), these were immunohistochemically validated to express high levels of HLA-DRA and CD74. Enhanced intercellular communication networks : Juvenile lesions showed active stromal-epithelial crosstalk mediated by ligand-receptor interactions such as PTN-SDC4 ($p < 0.001$) and MDK-NCL ($p = 0.003$), suggesting a role in tumor progression. CONCLUSIONS: Our findings reveal molecular features that may underlie the rapid growth of juvenile giant fibroepithelial tumors. The data point to hormone-independent proliferative programs and immune-modulating fibroblast subtypes as key contributors. These insights broaden our understanding of tumor biology and may guide the development of targeted, less invasive treatments for young patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12830078/

Record 116

pmcid: PMC12969900

pmid: 41559616

doi: 10.1186/s12884-025-08199-3

title: Somatic sensations throughout the pregnancy: their time course along with gestational weeks and relation to maternal-fetal attachment.

authors

- Tanaka Y
- Araki T
- Fukushima H

journal: BMC pregnancy and childbirth

journalAbbr: BMC Pregnancy Childbirth

year: 2026

pubdate: 2026 Jan 21

volume: 26

issue: 1

pages:

abstract: BACKGROUND: Pregnancy is responsible for a wide range of changes in a woman's somatic sensations, which may significantly impact a mother's attachment to the fetus. The present cross-sectional study aimed to investigate the transition of pregnant women's somatic sensations from early to late pregnancy and the relationship between maternal somatic sensations and attachment to the fetus. METHODS: Through a web-based questionnaire survey of first-time pregnant women, we measured their somatic sensations of pregnancy (SSoP), maternal-fetal attachment, and other indices. We conducted factor analysis on the SSoP to identify the factors within the SSoP involved. On this basis, we examined whether differences in gestational week (GWs) and factors affected the SSoP scores. Hierarchical multiple regression analysis was used to examine whether the SSoP factors were associated with maternal-fetal attachment. RESULTS: The factor analysis revealed that the SSoP were divided into seven factors according to the type of physical symptoms and the concerned body part, with each factor showing different patterns with the progression of GWs. Among the SSoP factors, the sensitivity to the urological system predicted a higher attachment total score after controlling for the effect of other indices. CONCLUSIONS: The findings accentuate the manifold changes in the somatic sensations experienced by pregnant women throughout the GWs and imply that somatic sensations are one of the significant factors for the development of maternal-fetal attachment. This comprehensive evaluation of somatic sensations in pregnant women has considerable implications for the promotion of their physical and psychological well-being.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12969900/

Record 117

pmcid: PMC13575667

pmid: 42746242

doi: 10.7759/cureus.114587

title: Spironolactone-Induced Bilateral Gynecomastia in a Patient With Comorbid Conditions.

authors

- Brahmbhatt SV
- Jain D

journal: Cureus

journalAbbr: Cureus

year: 2026

pubdate: 2026 Aug

volume: 18

issue: 8

pages: e114587

abstract: Gynecomastia is a benign proliferation of male breast glandular tissue caused by an imbalance between estrogen and androgen activity. Spironolactone, a potassium-sparing diuretic commonly prescribed for edema and ascites in chronic liver disease, is a recognized cause of drug-induced gynecomastia because of its antiandrogenic effects. We report the case of a man in his 50s with chronic liver disease and type 2 diabetes mellitus who presented with bilateral breast swelling and mild tenderness of one month's duration. Clinical examination revealed bilateral soft glandular enlargement without nipple discharge, a hard mass, or axillary lymphadenopathy. Breast ultrasonography demonstrated diffuse fibroglandular proliferation with subareolar thickening consistent with bilateral gynecomastia. Laboratory investigations showed mildly reduced serum testosterone with relative estrogen predominance, while prolactin, luteinizing hormone, follicle-stimulating hormone, and thyroid function tests were within normal limits. Chronic liver disease likely contributed to hormonal imbalance through impaired estrogen metabolism and altered sex hormone regulation, thereby increasing susceptibility to spironolactone-induced gynecomastia. Spironolactone therapy was discontinued, and conservative management with clinical monitoring was advised. The patient showed gradual symptomatic improvement during follow-up. This case highlights the importance of recognizing medication-induced endocrine adverse effects, particularly in patients with chronic systemic illnesses such as hepatic dysfunction.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13575667/

Record 118

pmcid: PMC13331895

pmid: 42397550

doi: 10.1007/s00520-026-10955-w

title: Symptoms experienced during the first 4 months of chemotherapy administration: a longitudinal cohort study in an Australian cancer treatment unit.

authors

- Alexander KE
- Ogle T
- Scanlon B
- Farley MJ
- Bradford N
- Pitt E
- Hoberg H
- Herron E
- Doyle L
- Abraham R
- Bentley M
- Inglis PL
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- Lane S
- Yates PM

journal: Supportive care in cancer : official journal of the Multinational Association of Supportive Care in Cancer

journalAbbr: Support Care Cancer

year: 2026

pubdate: 2026 Jul 3

volume: 34

issue: 7

pages:

abstract: PURPOSE: Individuals undergoing chemotherapy experience a range of symptoms that can significantly impact their quality of life and treatment outcomes; the prevalence and severity of which can change across their treatment journey. This study aimed to identify and describe the longitudinal symptom experience during the first four months of chemotherapy administration across a range of cancer diagnoses. METHODS: A prospective longitudinal cohort study was conducted at a cancer centre in Queensland, Australia. Monthly surveys assessed symptoms, distress, and quality of life across 4 months of chemotherapy. RESULTS: A total of 252 participants completed baseline surveys (mean age 61 ± 12.3 years; 65% female; 40% breast cancer). Participants reported a mean of 22.5 ± 9.0 symptoms at baseline, with a statistically significant reduction in symptoms throughout the study period 21.9 ± 9.2 symptoms (p = 0.002). Fatigue (93%), insomnia (80%), and pain (72%) were most common at baseline and remained highly prevalent throughout the time period. Severe fatigue (25%), pain (15%), and constipation (15%) reduced over time, while peripheral neuropathy and decreased sexual interest increased. CONCLUSION: This study has produced insights into the high number of symptoms experienced during the initial four months of chemotherapy. Given the impact of symptoms on treatment outcomes, there is a critical need for the development of appropriate assessment tools capable of capturing this broad range of patient symptom experiences. IMPLICATIONS FOR CANCER SURVIVORS: Understanding symptom patterns over time can support early identification of high-risk patients, guide personalised supportive care strategies and optimise treatment tolerance and quality of life during and beyond chemotherapy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13331895/

Record 119

pmcid: PMC13247343

pmid: 41697992

doi: 10.1093/jnci/djag039

title: Systematic review of patient-reported outcome item libraries in cancer research: an EORTC Quality of Life Group study.

authors

- Peacock R
- Bedding C
- Pacheco B
- Hamzeh H
- Piccinin C
- Gilbert A

journal: Journal of the National Cancer Institute

journalAbbr: J Natl Cancer Inst

year: 2026

pubdate: 2026 Jun 1

volume: 118

issue: 6

pages: 971-996

abstract: BACKGROUND: Patient-reported outcome (PRO) item libraries support flexible PRO assessment in cancer research by facilitating the development of customized item lists. However, little is known about the use of item lists in clinical research. This systematic review addresses this by assessing utilization of PRO item libraries and lists in oncology research. METHODS: A systematic review of MEDLINE, Embase, and CINAHL identified cancer studies using PRO item libraries to develop item lists, regardless of study design, published between October 2021 and September 2025. Key features of item library usage were extracted and analyzed descriptively. RESULTS: A total of 78 studies were included (25 trials and feasibility, 53 observational). The Patient-Reported Outcomes version of the Common Terminology Criteria for Adverse Events system was frequently used (49 of 78), and symptom assessment was the most common application (63 of 78). Item lists were implemented across different settings including novel treatments (19 of 78) and rare cancers (15 of 78). Most item lists were derived from a single PRO item library (72 of 78). In some studies, there was additional customization such as item wording changes (5 of 78) or addition of items adapted from the item library (9 of 78). Item selection methods included literature (32 of 78), patient involvement (8 of 78) and consultation with health-care professionals (11 of 78). Many studies (40 of 78) did not report methods used. CONCLUSIONS: PRO item libraries are increasingly used to create customized item lists in oncology research, primarily for symptom assessment. However, reporting practices for methods used are inconsistent, highlighting the need for standardized guidelines for reporting PRO item lists in clinical trials and routine care to improve transparency, reproducibility, and quality.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13247343/

Record 120

pmcid: PMC12966808

pmid: 41798434

doi: 10.7759/cureus.103002

title: T-cell Acute Lymphoblastic Leukemia Presenting With Bilateral Breast Masses.

authors

- Alsoukhni H
- Al-Mharat S
- Abu Darweesh I
- Alrousan A
- Bani Essa M

journal: Cureus

journalAbbr: Cureus

year: 2026

pubdate: 2026 Feb

volume: 18

issue: 2

pages: e103002

abstract: T-cell lymphoma involvement of the breast is rare and represents a diagnostic challenge. Here, we present a case of T-cell acute lymphoblastic leukemia (ALL) in a 20-year-old woman who presented in September 2023 with a history of bilateral breast enlargement associated with general weakness, weight loss, and a feeling of heat, which had lasted for about two months. She was examined clinically and found to have huge bilateral breast enlargement. A breast ultrasound (US) was performed and showed a diffuse abnormal heterogeneous parenchymal appearance of both breasts with multiple enlarged bilateral axillary lymph nodes. The patient was referred to the Military Cancer Center for breast core biopsy, which showed bilateral breast

involvement by T-cell lymphoblastic lymphoma. Subsequently, bone marrow examination revealed T-cell ALL. The patient was started on the augmented Berlin-Frankfurt-Münster protocol, achieved complete remission, and maintained it for two years after diagnosis. ALL in the breast is rare but should be considered especially in young patients presenting with sudden breast masses.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12966808/

Record 121

pmcid: PMC13384350

pmid: 42475644

doi: 10.2196/90567

title: Tailored Personas of Online Health Information-Seeking Behaviors Among Men With Prostate Cancer Receiving Androgen Deprivation Therapy: Qualitative Study.

authors

- Hou S
- Li X
- Qi Z
- Li K
- Chen L
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journal: Journal of medical Internet research

journalAbbr: J Med Internet Res

year: 2026

pubdate: 2026 Jul 20

volume: 28

issue:

pages: e90567

abstract: BACKGROUND: Patients with prostate cancer undergoing androgen deprivation therapy (ADT) must manage complex treatment side effects over extended periods outside the hospital, making online health information-seeking a key approach to self-management. However, individual differences in motivation, digital literacy, and psychosocial context significantly influence how patients seek and use online health information. The patient persona approach, which synthesizes individuals with similar behavioral patterns and needs into representative profiles, offers a practical method for capturing this heterogeneity and informing the design of tailored information support. OBJECTIVE: This study aimed to explore the differences in online health information-seeking behavior among patients with prostate cancer undergoing ADT and construct patient personas to characterize distinct patterns of online health information-seeking experiences and related information support needs. METHODS: A qualitative descriptive study was conducted in a tertiary hospital in Zhejiang Province, China, from July to October 2025. Purposive sampling was used to recruit patients receiving ADT. Semistructured interviews were conducted, and data were analyzed using inductive content analysis to derive codes, subcategories, and 4 core categories. Participants were systematically compared across these categories and grouped into personas based on recurring patterns. The personas were then presented with illustrative portraits. RESULTS: A total of 20 participants were included. Four core categories were identified for persona construction: motivation, information access preferences, barriers, and needs. The personas were categorized as follows: patients demonstrating proactive information management and high health literacy, patients exhibiting avoidant information-seeking driven by latent anxiety, patients engaging in family-doctor-mediated information-seeking, and patients displaying dependent and passive information-seeking behavior. CONCLUSIONS: Online health information-seeking behavior among patients with prostate cancer receiving

ADT is highly heterogeneous and shaped by individual capability, emotional responses, and sociocultural factors. The identified personas may provide a preliminary and practice-oriented framework for developing more differentiated, culturally sensitive, and patient-centered online health information support strategies for patients receiving long-term ADT.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13384350/

Record 122

pmcid: PMC13008910

pmid: 41883685

doi: 10.3389/fped.2026.1765982

title: Tamoxifen therapy in juvenile gigantomastia associated with PASH: a case report.

authors

- Coslovich S
- Bortul M
- Zanconati F
- Tornese G

journal: Frontiers in pediatrics

journalAbbr: Front Pediatr

year: 2026

pubdate: 2026

volume: 14

issue:

pages: 1765982

abstract: Pseudoangiomatous stromal hyperplasia (PASH) is a rare benign breast lesion in children. We report a 10-year-old girl with hormonally driven gigantomastia and recurrent mastitis, interpreted as a secondary inflammatory complication of rapid breast enlargement. Treatment with oral tamoxifen led to arrest of breast growth and resolution of inflammatory symptoms. This case supports tamoxifen as a potential non-surgical option for hormonally driven pediatric gigantomastia associated with PASH.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13008910/

Record 123

pmcid: PMC13607601

pmid: 42796107

doi: 10.3390/jcm15187334

title: Technique-Driven and Patient-Specific Variations in Complications of Mastectomy with Immediate Breast Reconstruction: An Evidence-Based Review.

authors

- Coyle L
- Odoom G
- Park YJ
- Siddiqui A
- Ricci JA
- Tanna N

journal: Journal of clinical medicine

journalAbbr: J Clin Med

year: 2026

pubdate: 2026 Sep 21

volume: 15

issue: 18

pages:

abstract: Mastectomy with immediate breast reconstruction (IBR) has become increasingly common, driven by advances in mastectomy techniques and the growing number of autologous, implant-based, and hybrid reconstructive options. The expansion of reconstructive strategies has introduced greater complexity in surgical planning and outcomes. Postoperative outcomes are influenced not only by mastectomy-related complications but also by complications specific to each reconstructive technique. Consequently, each combination of mastectomy and reconstructive modality carries unique benefits, limitations, and complication profiles that must be evaluated in the context of patient-specific factors, such as comorbidities, oncologic treatment, and reconstructive goals. This narrative review aims to characterize complication profiles associated with mastectomy and IBR to support improved perioperative management and individualized surgical care. To enhance clinical utility, this review includes representative clinical images that demonstrate variability in complication presentation across surgical approaches and patient contexts. This review presents a framework for understanding the factors that influence patient candidacy and outcomes in the setting of mastectomy with IBR, thereby supporting improved risk stratification and tailored treatment algorithms.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13607601/

Record 124

pmcid: PMC12865859

pmid: 41640900

doi: 10.7759/cureus.100644

title: Temporal Patterns of Adverse Events Associated With Selective Serotonin Reuptake Inhibitors: A Global Pharmacovigilance Analysis of Early-Onset Versus Late-Onset Toxicity.

authors

- Chan ACY

journal: Cureus

journalAbbr: Cureus

year: 2026

pubdate: 2026 Jan

volume: 18

issue: 1

pages: e100644

abstract: Background Selective serotonin reuptake inhibitors (SSRIs) are commonly used as first-line antidepressant medications, but adverse events (AEs) remain a common reason for treatment discontinuation. Understanding when these AEs occur can help improve patient care and treatment adherence. Objective This study aims to explore the timing of AEs associated with six commonly used SSRIs: sertraline, fluoxetine, fluvoxamine, paroxetine, citalopram, and escitalopram using global pharmacovigilance data, with a focus on early- versus late-onset profiles. Methods This study analyzed Individual Case Safety Reports (ICSRs) from VigiBase, the World Health Organization's global safety database. Reports were included if an SSRI was the suspected drug and time-to-onset (TTO) data were available. AEs with at least 10 reported TTOs were grouped as early-onset (TTO ≤28 days) or late-onset (TTO >28 days). Results A total of 1,428 AEs met the inclusion

criteria. Of these, 914 (64%) were early onset, including nausea (median TTO: 1 day), insomnia and dizziness (2 days), and sexual dysfunction (16.5 days). Late-onset AEs, 514 (36%), included weight gain (31 days), hyperhidrosis (76.5 days), diabetes mellitus (151 days), and osteoporosis (959.5 days). Early AEs were mostly gastrointestinal, neurological, or activation-related; late AEs were largely metabolic or endocrine. Conclusions SSRIs show distinct temporal AE patterns. Early-onset symptoms require timely management to improve tolerability, while late-onset effects highlight the need for ongoing monitoring. These findings can inform personalized monitoring strategies and guide patient counseling to support safer long-term SSRI use.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12865859/

Record 125

pmcid: PMC13187599

pmid: 42105390

doi: 10.1016/j.psj.2026.107016

title: The effect of housing system and broiler breed on meat quality(1).

authors

- Caceres JC
- Sobreira CH
- Munmun AR
- Sigmon CS
- He-DeMontaron AD
- Pullin AN
- Grimes J
- Walker LL
- Schilling MW
- Campbell YL

journal: Poultry science

journalAbbr: Poult Sci

year: 2026

pubdate: 2026 Aug

volume: 105

issue: 8

pages: 107016

abstract: This study investigated the effects of the silvopastoral (SP) rearing system compared with a conventional (CV) indoor system, along with the influence of broiler breed (fast-growing, FAST vs. slow-growing, SLOW), on carcass traits (yields), meat quality, myopathy incidence, fatty acid profile, oxidative stability (TBARS), and sensory attributes. A 2 × 2 factorial design was conducted, using 250 birds per treatment, with 5 replicate pens (50 birds/pen). For data collection, 10 birds were selected from each pen for processing, giving a total of 5 replicates and 50 samples per treatment. FAST consistently exhibited superior carcass and breast yields, while SLOW had minimal breast muscle myopathies, regardless of the system. Several physicochemical traits were primarily influenced by the breed, with SLOW's breast exhibiting a greater content of connective tissue and shear force. In comparison, FAST exhibited greater moisture content. The housing system influenced lipid composition and oxidative stability, with SP meat exhibiting lower saturated fatty acid content than CV (40.16% vs. 42.04%) and reduced TBARS values (1.50 vs. 1.59 mg MDA/kg). However, the effect of SP on FAST was limited, as cooking loss did not differ between SP and CV systems (30.91% vs. 32.76%). Sensory results showed minimal differences; meat from CV birds was rated slightly juicier, while SLOW's meat was described as chewier and gummier. Overall, trade-offs were identified between yield performance and meat

quality; meat from fast-growing birds had greater production efficiency but exhibited a greater incidence of muscular abnormalities. SP production demonstrated the potential to enhance certain meat quality traits without compromising sensory acceptance, albeit at a lower yield expense.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13187599/

Record 126

pmcid: PMC13425464

pmid: 42540206

doi: 10.1155/rrp/7490766

title: The Gems and Pitfalls of Inflammatory Breast Disease: An Illustrated Review of Mastitis. What the Radiologist Should Know.

authors

- Boldrini C
- Bottalico M
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- Belli P

journal: Radiology research and practice

journalAbbr: Radiol Res Pract

year: 2026

pubdate: 2026

volume: 2026

issue:

pages: 7490766

abstract: Mastitis must be considered as a heterogeneous group of inflammatory breast conditions with varying aetiologies, clinical presentations, and management. The primary objective of this pictorial review is to clarify the role of imaging in diagnosis of mastitis, and, above all, in the differential diagnosis of breast cancer (BC), particularly inflammatory cancer. The main literature regarding mastitis published in the PubMed database over the past 18 years was examined. Mastitis is classified into infectious and noninfectious forms, including inflammatory conditions associated with or overlapping with cancer. Infectious mastitis, more common in women of childbearing age and during breastfeeding, is generally caused by bacteria (especially Staphylococcus aureus). Ultrasound is the first-line examination to confirm the inflammatory process and identify any abscess clusters, while mammography has a limited role in the acute phase; MRI may be useful in complex or atypical cases. Noninfectious mastitis includes forms such as periductal mastitis, idiopathic granulomatous mastitis, and inflammatory reactions to foreign bodies or trauma. They can clinically and radiologically mimic cancer. Imaging often shows skin thickening, focal or diffuse parenchymal changes, and irregular hypoechoic areas, sometimes requiring biopsy for a definitive diagnosis. Inflammatory BC is a rare but aggressive entity that can present with clinical signs similar to mastitis. Failure to respond to antibiotic therapy, persistent clinical signs, and certain imaging patterns (especially MRI) should always raise suspicion of cancer and prompt further histological investigations. In conclusion, in the context of inflammatory breast pathology, the radiologist plays a key role in recognizing signs suggestive of benign or malignant disease, ensuring the appropriate management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13425464/

Record 127

pmcid: PMC12903031

pmid: 41694167

doi: 10.7759/cureus.103467

title: The Role and Feasibility of Automated Breast Ultrasound in the Evaluation of Male Breast Disease: Workflow Efficiency and Imaging Spectrum.

authors

- Eren A
- Karatay E
- Durur-Subasi I

journal: Cureus

journalAbbr: Cureus

year: 2026

pubdate: 2026 Feb

volume: 18

issue: 2

pages: e103467

abstract: Traditional hand-held ultrasonography (HHUS) suffers from inherent limitations, including operator dependency, lack of reproducibility, and long acquisition times in busy clinical settings. Automated breast ultrasound (ABUS) addresses these drawbacks by providing standardized, three-dimensional, and operator-independent imaging that separates acquisition from interpretation. While the diagnostic performance of ABUS has been extensively validated for supplemental screening in females with dense breast tissue, its specific application in the male population remains an unexplored frontier in the literature. Objective The objective of this study was to evaluate the diagnostic value, workflow advantages, and imaging spectrum of ABUS in male breast diseases based on a large-scale, single-center experience. Methods This retrospective study included 85 male patients (mean age: 36.6 ± 13.7 years) who underwent ABUS between April 2023 and February 2025. Inclusion criteria focused on symptomatic patients and those requiring high-risk screening or follow-up. Recall rates, complementary imaging frequency, and the Breast Imaging Reporting and Data System or BI-RADS categories were analyzed. Results Breast pain (53 cases, 62.4%) was the most common symptom, and gynecomastia (29 cases, 34.1%) was the most frequent finding. The recall rate for secondary HHUS was 1.2% (1 case). Three biopsies revealed one lipoma, one invasive ductal carcinoma, and one papillary carcinoma. Conclusion ABUS provides a standardized, reproducible evaluation of male breast diseases with minimal recall and optimized workflow efficiency. It serves as a promising adjunct to standard imaging protocols, particularly in younger males where avoiding radiation is preferable, and offers a unique anatomical plane (C-plane).

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12903031/

Record 128

pmcid: PMC12932513

pmid: 41756513

doi: 10.3389/fcvm.2026.1710507

title: Thromboembolic events with olanzapine: a systematic review integrating meta-analysis and FAERS database.

authors

- Zhang X
- Sun S
- Fan X
- Guo S
- Li W
- Wang C

- Xu J
- Chen C
- Jin H

journal: Frontiers in cardiovascular medicine

journalAbbr: Front Cardiovasc Med

year: 2026

pubdate: 2026

volume: 13

issue:

pages: 1710507

abstract: BACKGROUND: Olanzapine is an atypical antipsychotic used to treat schizophrenia and manic episodes. Its potential thromboembolic risk has been reported, but the evidence remains controversial. This study aimed to comprehensively evaluate the association between olanzapine and pulmonary embolism (PE) and venous thromboembolism (VTE). METHODS: This study combined meta-analysis and signal mining from the FDA Adverse Event Reporting System (FAERS) database to assess the association between olanzapine and pulmonary embolism (PE) and venous thromboembolism (VTE). RESULTS: From 55,905 olanzapine-related adverse event reports in the Faers database, 1,233 significant signals were identified, including serious adverse events not fully documented on the drug label, such as pulmonary embolism and venous embolism. A meta-analysis of eight studies showed that olanzapine use significantly increased the risk of VTE and pulmonary embolism (OR = 2.07, 95% CI: 1.37-3.14, P = 0.0006). CONCLUSION: These findings suggest that olanzapine is associated with an increased risk of thromboembolic events; therefore, enhanced clinical surveillance and further investigation into its safety are necessary. SYSTEMATIC REVIEW REGISTRATION: https://www.crd.york.ac.uk/prospero/, identifier CRD420251003254.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12932513/

Record 129

pmcid: PMC13623785

pmid: 42811175

doi: 10.1007/s00431-026-07454-9

title: Trajectory-based analysis of growth in congenital adrenal hyperplasia: therapeutic implications for final height preservation.

authors

- Gawlik Z
- Gawlik J
- Tyrawa K
- Starzyk J
- Januś D

journal: European journal of pediatrics

journalAbbr: Eur J Pediatr

year: 2026

pubdate: 2026 Sep 30

volume: 185

issue: 10

pages:

abstract: UNLABELLED: Congenital adrenal hyperplasia (CAH) is an autosomal recessive genetic disorder resulting in a dysfunction of the steroidogenesis pathway and glucocorticoid deficiency. Proper care demands not only preventing acute complications but also optimizing growth and sexual development. This study was set out to analyze final height metrics and the factors affecting them as a long-term outcome measure in CAH patients. This observational retrospective study evaluated almost 3000 visits of 63 CAH patients treated in one center between 1996 and 2025, who reached their near-final height (nFH). The population was divided into the salt-wasting (SW, n = 45) and non-SW groups (n = 18). The main outcome measures were nFH, corrected nFH (c-nFH), and their standard deviation scores (SDSs). They were associated with numerous factors, including glucocorticoid doses, bone age (BA), height velocity (HV), occurrence of puberty, body mass index, and adiposity rebound. c-nFH SDS was compromised in both the SW (- 0.99 ± 0.87) and non-SW patients (- 0.57 ± 0.71) (p = 0.053). HV trajectories pointed to a flattened pubertal growth spurt in both populations. Maximum BA advancement (BAA) and peak ΔBAA were lower in SW than in non-SW patients, while age at peak ΔBAA and age at maximum BAA were comparable between groups. Both longitudinal and cross-sectional regression models showed negative interactions between hydrocortisone doses and height outcomes. CONCLUSION: We observed that CAH patients tend to have compromised final height, with the SW-CAH population presenting with most compromised growth. Higher hydrocortisone doses were associated with worse outcomes. The analysis of BA and HV trajectories seems to suggest a period demanding intensified care. WHAT IS KNOWN: • CAH and long-term glucocorticoid therapy may impair growth and final height, particularly in salt-wasting forms, with bone age advancement regarded as a major determinant of height outcome. • Higher hydrocortisone exposure has been associated with poorer growth outcomes in CAH patients. WHAT IS NEW: • Longitudinal analysis of 2949 visits identified a shared critical period between 6 and 10 years of age characterized by accelerated bone age advancement and altered height velocity trajectories requiring intensified disease control. • Early optimization of androgen suppression and consideration of aromatase inhibitor therapy may improve growth outcomes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13623785/

Record 130

pmcid: PMC12992300

pmid: 41853244

doi: 10.3389/fped.2026.1769902

title: Trends in central precocious puberty incidence in Japan during the COVID-19 pandemic.

authors

- Komatsu Y
- Kikuchi N
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- Ohsugi K
- Shiga K

journal: Frontiers in pediatrics

journalAbbr: Front Pediatr

year: 2026

pubdate: 2026

volume: 14

issue:

pages: 1769902

abstract: BACKGROUND: The incidence of central precocious puberty (CPP) has reportedly increased worldwide during the coronavirus disease 2019 (COVID-19) pandemic; however, multicenter data from Japan remain limited. This study aimed to evaluate temporal changes in the incidence and clinical characteristics of CPP before, during, and after the pandemic. METHODS: We conducted a multicenter retrospective observational study across four pediatric endocrinology centers in Kanagawa, Japan. Newly diagnosed CPP

cases from 2018 to 2023 were categorized into three periods: pre-pandemic (2018-2019), pandemic (2020-2021), and post-pandemic (2022-2023). Incidence rate ratios (IRRs) were calculated using quasi-Poisson regression, with population size included as an offset. Clinical characteristics-including age at diagnosis, bone age, degree of overweight, and hormone profiles-were compared across periods using the Kruskal-Wallis test. RESULTS: A total of 118 children (94 girls and 24 boys) were diagnosed with CPP during the study period. Among girls, CPP incidence increased significantly during the pandemic compared with the pre-pandemic period [IRR 2.47; 95% confidence interval (CI): 1.29-5.03]. In boys, incidence also increased with a statistically significant IRR; however, the estimate was accompanied by wide confidence intervals owing to the small number of cases. Elevated incidence rates in girls persisted into the post-pandemic period. No significant differences were observed across periods in age at diagnosis, degree of bone age advancement, degree of overweight, or basal and stimulated hormone levels. Nevertheless, the cohort consistently exhibited higher degrees of overweight compared with national reference values. CONCLUSIONS: This multicenter study demonstrates a significant increase in CPP incidence among girls during the COVID-19 pandemic in Japan, with sustained elevation in the post-pandemic period. Although clinical characteristics remained largely unchanged, the consistently higher degree of overweight underscores the need to consider lifestyle and environmental factors that may have been exacerbated during the pandemic. Ongoing surveillance and reevaluation of CPP diagnostic criteria may be warranted to address emerging epidemiological trends.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12992300/

Record 131

pmcid: PMC13503043

pmid: 42639324

doi: 10.7759/cureus.113374

title: Unmasking Ehlers-Danlos Syndrome Through Reduction Mammaplasty: An Illustrative Case and a Systematic Literature Review.

authors

- Gibbons V
- Batubo V
- Woodberry KM

journal: Cureus

journalAbbr: Cureus

year: 2026

pubdate: 2026 Jul

volume: 18

issue: 7

pages: e113374

abstract: Ehlers-Danlos syndrome (EDS) is characterized by joint hypermobility, skin hyperextensibility, and tissue fragility, which may worsen symptomatic macromastia. Concerns about elevated surgical risk have historically discouraged elective procedures in EDS patients; however, postoperative complications may themselves prompt an EDS diagnosis, as illustrated by our case. This relationship prompted the conduct of a systematic review examining outcomes of reduction mammaplasty in EDS. A 37-year-old woman underwent bilateral inferior pedicle reduction mammaplasty for symptomatic mammary hyperplasia. Persistent intraoperative oozing resulted in postoperative anemia (hemoglobin decline from 13.3 g/dL to 10.4 g/dL). Additional complications included delayed wound healing, hypertrophic scarring, and transient left upper extremity weakness. Hematologic evaluation was unremarkable. Clinical assessment revealed joint hypermobility (Beighton score ≥5/9), positive Walker and Steinberg signs, skin hyperextensibility, easy bruising, and menorrhagia, leading to a clinical diagnosis of hypermobile EDS. A systematic review of PubMed, Scopus, and Google Scholar (December 1950 to June 2025) was performed and included all study designs. Extracted data included demographics, interventions, complications, revision rates, and satisfaction. Two independent

reviewers assessed quality using Joanna Briggs Institute (JBI) critical appraisal tools. Of the 115 identified records, the following four studies met the inclusion criteria: one case report, one case series, one retrospective cohort study, and one clinical reference. All reported cases involved patients with pre-existing EDS diagnoses and demonstrated successful outcomes with symptomatic improvement and few complications. The cohort study consisted of multiple elective procedures, including reduction mammaplasty, and reported fewer complications in EDS patients compared to matched controls, but did not report outcomes specific to the reduction mammaplasty group. The clinical reference emphasized meticulous operative planning. This systematic review with an illustrative case underscores the need for further research and updated clinical guidance. Although existing literature largely challenges the view of EDS as a surgical contraindication, our case demonstrates complications consistent with known EDS-related risks and suggests a potential role for plastic surgeons in identifying undiagnosed EDS.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13503043/

Record 132

pmcid: PMC13274900

pmid: 42308183

doi: 10.1371/journal.pmen.0000614

title: Unveiling the hidden burden: Common mental disorders among women attending antenatal care in selected health facilities of Lusaka, Zambia.

authors

- Jacobs C
- Nkhowani J
- Ngosa D
- Shanaube K
- Sandøy IF

journal: PLOS mental health

journalAbbr: PLOS Ment Health

year: 2026

pubdate: 2026

volume: 3

issue: 6

pages: e0000614

abstract: Mental health disorders disproportionately affect women during pregnancy and postpartum, with high prevalence in low-income countries. Mental health disorders during the antenatal period can lead to serious complications for both mother and child, including preterm birth, developmental issues and postpartum depression. However, mental health screening remains neglected during the antenatal period in resource-limited settings. We investigated the prevalence and determinants of common mental disorders (CMD) among women attending antenatal care (ANC) in selected health facilities of Lusaka, Zambia. A cross-sectional study was conducted between 26th April and 6th July 2024 in two purposively selected health facilities in low socioeconomic areas of Lusaka. A systematic sample of pregnant women in the third-trimester attending ANC were invited to participate. Common mental disorders were defined using a cut-off score of ≥8 on the WHO Self-Reporting Questionnaire (SRQ)-20, administered through face-to-face interviews. Logistic regression models were employed to identify determinants of common mental disorders. Stata 18 SE was used for analysis. We included 331 participants who were predominantly young adults (50.5% aged 20-29), married/cohabiting (69%), and almost a quarter (23.9%) living with HIV. Most had unplanned pregnancies (58.6%), experienced pregnancy-related body changes (73.1%) and had delivery concerns (68.6%). The study found a high prevalence of common mental disorders (65.8%). Participants who were less than 20 years, primary level education, had history of complications during previous pregnancy, were worried about the birth process and

reported unpleasant body changes due to pregnancy exhibited higher prevalence for common mental disorders. On the other hand, women with medium or high social connectedness had lower odds of common mental disorders. The high burden of common mental health disorders among pregnant women in Lusaka, Zambia, underscores the importance of integrating mental health screening and support into routine antenatal care, with targeted attention to high-risk groups such as adolescents.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13274900/

Record 133

pmcid: PMC13108661

pmid: 42040527

doi: 10.1097/GOX.0000000000007660

title: Vertical Incision for Cosmetic Reduction of Giant Benign Breast Masses: A Case Report and Literature Review.

authors

- Liu R
- Huang J
- Yang Z
- Huang Y
- Wang S
- Lei Z
- Yang Z

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2026

pubdate: 2026 Apr

volume: 14

issue: 4

pages: e7660

abstract: BACKGROUND: Giant benign breast masses cause unilateral or bilateral breast enlargement, which can lead to discomfort, including swelling and pain, and adversely affect appearance and daily activities. The surgical management of giant benign breast masses presents challenges in achieving a balance between therapeutic efficacy and cosmetic outcomes. METHODS: A comprehensive analysis of the historical development and impact of modifications in surgical techniques for breast reduction was conducted through a literature review. Meanwhile, the surgical approach and perioperative outcomes associated with a giant hamartoma in the left breast were documented. RESULTS: Following the excision of the giant benign breast mass, the residual mammary globules were selectively preserved, despite the presence of excess skin. Consequently, a vertical incision technique was used for breast reduction, effectively excising the excess skin and minimizing scarring. Furthermore, the sensation and function of the nipple-areola complex were more effectively preserved, resulting in an improved and more aesthetically pleasing breast contour. CONCLUSIONS: A vertical incision for giant benign breast masses, combined with an individualized surgical approach, allows for better preservation of nipple-areola complex sensation and promotes a favorable and long-lasting breast shape, indicating significant value for this surgical design in patients with giant benign breast masses in general surgery.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13108661/

Record 134

pmcid: PMC13151937

pmid: 42111331

doi: 10.2147/IJWH.S596549

title: When Lifelong Anosmia Reveals Hypogonadism: A Case of Kallmann Syndrome with Primary Amenorrhea.

authors

- Lubis JYG
- Susilo AFP
- Anwar R
- Rachmawati A

journal: International journal of women's health

journalAbbr: Int J Womens Health

year: 2026

pubdate: 2026

volume: 18

issue:

pages: 596549

abstract: BACKGROUND: Kallmann syndrome (KS) is a rare genetic disorder characterized by hypogonadotropic hypogonadism and anosmia. It is often underdiagnosed in females due to subtle clinical presentation. This can lead to delays in accurate diagnosis and proper long-term management, particularly in women presenting with amenorrhea and uterine hypoplasia. CASE ILLUSTRATION: A 25-year-old nulligravid woman presented with lifelong absence of spontaneous menstruation, anosmia, and partial pubertal development. She was initially misdiagnosed with secondary amenorrhea after a positive progesterone withdrawal test, but later recognized as having primary amenorrhea. Examination revealed Tanner stage M3P3, uterine and ovarian hypoplasia on transvaginal ultrasonography, low estradiol with suppressed gonadotropins, normal 46,XX karyotype, and bilateral absence of the olfactory bulbs on brain MRI, confirming Kallmann syndrome. Dual-energy X-ray absorptiometry demonstrated reduced bone mass consistent with chronic hypoestrogenism. She was started on combined estrogen-progestin therapy with calcium and vitamin D supplementation for estrogen priming and bone protection, with fertility-oriented management and baseline psychological screening integrated into care. DISCUSSION: This case illustrates the diagnostic challenges of Kallmann syndrome in women with partial pubertal development. Insufficient estrogen exposure may allow secondary sexual characteristic development while failing to achieve uterine maturation and spontaneous menstruation. Chronic hypoestrogenism contributes not only to reproductive tract hypoplasia but also poses long-term risks to bone, cardiovascular, metabolic, and mental health, underscoring the importance of comprehensive evaluation and management. CONCLUSION: Kallmann Syndrome should be suspected in any female patient with hypogonadotropic hypogonadism and anosmia. Withdrawal bleeding should not be mistaken for true menarche, as this may obscure the diagnosis. Early diagnosis is essential to optimize hormonal replacement, preserve fertility potential, and improve long-term systemic health outcomes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13151937/

Record 135

pmcid: PMC13111392

pmid: 42052070

doi: 10.3389/fpsyg.2026.1797201

title: Women's and men's perceptions of (not) wearing a bra in public.

authors

- Prokop P
- Tomanová Čergeťová I

- Balcerčík J

journal: Frontiers in psychology

journalAbbr: Front Psychol

year: 2026

pubdate: 2026

volume: 17

issue:

pages: 1797201

abstract: INTRODUCTION: Human females often use clothing as a strategy for self-promotion to enhance their physical attractiveness to potential mates. The conscious concealment or revelation of intimate body parts can be used to solicit attention; however, little empirical research exists on this topic. METHODS: We investigated the factors influencing the decision to wear a bra using self-reports from Slovak women (N = 409) in public and private settings. Furthermore, using visual stimuli, we examined how male (N = 277) and female (N = 158) raters perceived the attractiveness and faithfulness of women with and without a bra. RESULTS: We found that being braless was significantly less frequent in public than in private settings. The decision was negatively associated with the fear of sexual harassment and positively associated with smaller, firmer breasts, but not with unrestricted sociosexuality. The braless state was consistently perceived by both men and women as more attractive, yet also as less faithful. Males with a stronger preference for large breasts found the braless state more attractive than those with a lower preference. Finally, male intentions to sexually harass females, as well as an unrestricted sociosexuality in male raters, were positively correlated with the perception of the braless state as a cue of infidelity. DISCUSSION: Our results suggest that bra-wearing represents a strategic trade-off in self-promotion: while it may dampen sexual attractiveness, it simultaneously protects the wearer from being perceived as infidel.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13111392/

Record 136

pmcid: PMC11729124

pmid: 39126560

doi: 10.1007/s40618-024-02435-x

title: "Management of andrological disorders from childhood and adolescence to transition age: guidelines from the Italian Society of Andrology and Sexual Medicine (SIAMS) in collaboration with the Italian Society for Pediatric Endocrinology and Diabetology (SIEDP)-Part-1".

authors

- Bonomi M
- Cangiano B
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- Garolla A
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- Lanfranco F
- Rastrelli G
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journal: Journal of endocrinological investigation

journalAbbr: J Endocrinol Invest

year: 2025

pubdate: 2025 Jan

volume: 48

issue: 1

pages: 1-22

abstract: PURPOSE: Andrological pathologies in the adulthood are often the results of conditions that originate during childhood and adolescence and sometimes even during gestation and neonatal period. Unfortunately, the reports in the literature concerning pediatric andrological diseases are scares and mainly concerning single issues. Furthermore, no shared position statement are so far available. METHODS: The Italian Society of Andrology and Sexual Medicine (SIAMS) commissioned an expert task force involving the Italian Society of Pediatric Endocrinology and Diabetology (SIEDP) to provide an updated guideline on the diagnosis and management of andrological disorders from childhood and adolescence to transition age. Derived recommendations were based on the grading of recommendations, assessment, development, and evaluation (GRADE) system. RESULTS: A literature search of articles in English for the term "varicoceles", "gynecomastia", "fertility preservation", "macroorchidism", "precocious puberty" and "pubertal delay" has been performed. Three major aspects for each considered disorder were assessed including diagnosis, clinical management, and treatment. Recommendations and suggestions have been provided for each of the mentioned andrological disorders. CONCLUSIONS: These are the first guidelines based on a multidisciplinary approach that involves important societies related to the field of andrological medicine from pediatric to transition and adult ages. This fruitful discussion allowed for a general agreement on several recommendations and suggestions to be reached, which can support all stakeholders in improving andrological and general health of the transitional age.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11729124/

Record 137

pmcid: PMC12424563

pmid: 40950542

doi: 10.47895/amp.vi0.10531

title: "They say it may cause cancer:" A Qualitative Exploration of Filipinos' Contraceptive Misconceptions and Primary Healthcare Interventions.

authors

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- Carandang RRZ

journal: Acta medica Philippina

journalAbbr: Acta Med Philipp

year: 2025

pubdate: 2025

volume: 59

issue: 11

pages: 8-17

abstract: BACKGROUND AND OBJECTIVE: Contraceptives are widely acknowledged for preventing unwanted pregnancies. However, there is a prevalent lack of awareness regarding contraceptives, leading to unaddressed misconceptions. This study aimed to identify common contraceptive misconceptions among men and women of reproductive age and explore how primary health workers address them. METHODS: A qualitative study was conducted in two phases within District 5, Manila City. Phase 1 comprised focused group discussions with men and women of reproductive age (n=60), while Phase 2 involved conducting in-depth interviews with primary healthcare providers (n=16). MAXQDA, a qualitative software, to organize and code the data, was utilized. RESULTS: Women of reproductive age reported several misconceptions about contraceptives, including concerns about adverse health effects, emotional and behavioral changes, perceived ineffectiveness, and cosmetic or bodily changes. For instance, they believed that contraceptives could lead to serious health complications, such as cancer, genital injury, and even death. Primary healthcare providers addressed these misconceptions through open dialogue during service delivery, particularly during prenatal and postpartum check-ups and infant immunizations. They utilized patient education strategies, including the teach-back method, and conducted community outreach and workshops on contraceptives and family planning, especially during Women's Month. CONCLUSION: Several misconceptions were identified among women of reproductive age regarding the proper use and safety of contraceptive methods, as well as misguided beliefs. In contrast, men did not exhibit any misconceptions about contraceptives, which warrants further investigation. Primary healthcare providers have taken a proactive approach to address this issue by offering comprehensive explanations and ensuring clear understanding between healthcare providers and women. Promoting contraceptive health literacy could help bridge the knowledge gap between men and women of reproductive age.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12424563/

Record 138

pmcid: PMC12496512

pmid: 40865509

doi: 10.1016/j.ijscr.2025.111804

title: A case report of male breast cancer treated with breast-conserving surgery.

authors

- Yoneyama K
- Nakagawa M
- Hara A

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2025

pubdate: 2025 Oct

volume: 135

issue:

pages: 111804

abstract: INTRODUCTION AND IMPORTANCE: Male breast cancer is a rare disease that accounts for less than 1 % of all breast cancer cases. The most common surgical treatment for male breast cancer is mastectomy, but breast-conserving surgery has been performed increasingly in recent years. This report describes a case of male breast cancer that was treated by breast-conserving surgery. CASE PRESENTATION: A 61-year-old man visited our hospital complaining of a painless mass in his left breast. Imaging revealed a well-defined cystic mass, aspiration of which revealed bloody fluid. Cytology was inconclusive, but the findings were suggestive of malignancy. A left lumpectomy was performed for diagnostic purposes. Postoperative pathology revealed invasive ductal carcinoma with a predominant intraductal component. The surgical margins were

negative. The patient declined adjuvant hormone therapy and radiotherapy and has been kept under follow-up in the year since surgery. There has been no recurrence during this time. CLINICAL DISCUSSION: As in women, the goals of treatment for breast cancer in men are oncological safety and satisfactory cosmetic results. In the absence of specific guidelines for the treatment of male breast cancer, mastectomy is currently the standard treatment. However, in recent years, breast-conserving surgery has been performed for tumors that are discovered early and do not invade the nipple and areola. If curability and safety can be guaranteed, breast-conserving surgery can be considered for male breast cancer. CONCLUSION: Breast-conserving surgery can be an effective treatment option in men with breast cancer.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12496512/

Record 139

pmcid: PMC12476806

pmid: 41025061

doi: 10.4329/wjr.v17.i9.110906

title: A compendium of male breast imaging: The road less traveled.

authors

- Singla V
- Bhatia H
- Garg D
- Bal A
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journal: World journal of radiology

journalAbbr: World J Radiol

year: 2025

pubdate: 2025 Sep 28

volume: 17

issue: 9

pages: 110906

abstract: Male breast disorders, though less prevalent, present unique diagnostic challenges that differ significantly from their female counterparts. While benign entities such as gynecomastia are predominant, the risk of underlying malignancy, often diagnosed at an advanced stage, highlights the need for a systematic, image-guided assessment. Ultrasound and mammography are the first-line complementary tools, with magnetic resonance imaging reserved for problem-solving. This review outlines the anatomical, pathological, and radiological nuances of the male breast, describing crucial red flag signs, sonographic pitfalls, and mammographic mimics that aid in distinguishing benign entities from sinister pathologies such as invasive ductal carcinoma. Given the increasing visibility of transgender individuals, this review also addresses imaging considerations and screening recommendations tailored to this population. By integrating clinical insights with radiologic imaging, this review offers a comprehensive approach to both common and not-so-common male breast lesions, with an emphasis on an algorithmic stepwise diagnostic approach.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12476806/

Record 140

pmcid: PMC11992803

pmid: 40217533

doi: 10.1186/s13002-025-00767-3

title: A comprehensive review on animals and their products used in traditional folk medicine in Ethiopia.

authors

- Assefa A
- Mesfin K
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journal: Journal of ethnobiology and ethnomedicine

journalAbbr: J Ethnobiol Ethnomed

year: 2025

pubdate: 2025 Apr 11

volume: 21

issue: 1

pages: 24

abstract: BACKGROUND: Zootherapy is the treatment of human and livestock ailments using medicines derived from animals and their products. The objective of this review paper was to compile and document ethnozoological knowledge on traditional medicinal animals used to treat various human and livestock ailments, along with the associated indigenous knowledge from different parts of Ethiopia. METHODS: Scientific databases were employed to conduct an exhaustive systematic search of published articles in the English language. A collection of 21 articles focused on animals and their by-products utilized in Ethiopian traditional medicine was compiled from accessible international online databases like Scopus, Web of Science, Google Scholar, PubMed, Research Gate, and various journal Web sites. RESULTS: According to the sources, a total 112 animal species used to treat 167 different ailments were identified. Among these, mammals are the most frequently used, followed by birds, arthropods, and reptiles. Most remedies are sourced from wild animals, with treatments for serious diseases, including HIV/AIDS and cancer, being reported. The most commonly used animal parts for medicine preparation are meat and fat, followed by internal organs, with cooking being the predominant preparation method. CONCLUSION: The medicinal use of animals is under threat due to illegal hunting and deforestation. The Ethiopian people possess rich indigenous knowledge of animal-based traditional medicine. To ensure the sustainable use of medicinal animals and support the development of modern medicine, further comprehensive ethnozoological research is urgently needed.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11992803/

Record 141

pmcid: PMC11776416

pmid: 39881782

doi: 10.2147/IMCRJ.S496803

title: A Dermatological Intervention of Gynecomastia in Young Asian Man with a History of Soy Product Consumption: A Case Report.

authors

- Widjajahakim R
- Widjajahakim RM
- Winaya KK

journal: International medical case reports journal

journalAbbr: Int Med Case Rep J

year: 2025

pubdate: 2025

volume: 18

issue:

pages: 173-179

abstract: Gynecomastia is an enlargement of the breasts in men, which is usually benign but can also manifest as a result of malignancies, and can be either unilateral or bilateral. Pseudogynecomastia is male breast enlargement due to excessive fat, while true gynecomastia is a proliferation of glandular tissue. Gynecomastia is common in infants, adolescents, and elderly men, with the most common cause is related to hormonal changes associated with aging. Medical intervention is usually required for true gynecomastia that has lasted more than a year. Here, we report a case of a 33-year-old Chinese-Indonesian decent with enlarged breasts. The patient reported frequent consumption of homemade soybean milk (1/2 to 1 liter daily) accompanied by bland chicken during his body-building regimen. The patient had discontinued the regimen and the diet for 2 years prior to the initial visit. During physical examination, both breasts were enlarged and had firm nodule, mobile, and attached at the sub-central areola mammae with a diameter of 8 cm. The patient was then diagnosed with true gynecomastia Simon degree 2A and Geschikter and Copeland type 3. As the gynecomastia had persisted longer than a year, a dermatological intervention was planned. Ultrasound-assisted liposuction was performed with a solid probe and glandular excision. Fat tissue was obtained, 130 mL from the right breast and 120 mL from the left breast. A dense 2.5 cm × 2.5 cm × 2 cm glandular tissue was obtained from both breasts. The patient was satisfied with the surgery outcome.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11776416/

Record 142

pmcid: PMC12085321

pmid: 40131719

doi: 10.1007/s40257-025-00935-x

title: A Guide to the Management of Hidradenitis Suppurativa in Pregnancy and Lactation.

authors

- Ghanshani R
- Lee K
- Crew AB
- Shi VY
- Hsiao JL

journal: American journal of clinical dermatology

journalAbbr: Am J Clin Dermatol

year: 2025

pubdate: 2025 May

volume: 26

issue: 3

pages: 345-360

abstract: Hidradenitis suppurativa is a chronic inflammatory condition characterized by recurrent abscesses, nodules, tunnels, and scarring. Fluctuations in disease activity are common during pregnancy, and more than half of women with hidradenitis suppurativa report experiencing post-partum flares. Both treatment efficacy and safety of the woman and fetus or infant must be considered when developing a treatment plan for pregnant and lactating women with hidradenitis suppurativa. Although certain commonly used hidradenitis suppurativa medications, such as tetracyclines and spironolactone, are contraindicated during pregnancy, there are still various medical therapies, including topicals, systemic antibiotics, metabolic modulators, and biologics, as well as procedural therapies that may be utilized during pregnancy. This paper aims to provide an updated evidence-based review of the management of hidradenitis suppurativa in pregnancy with an emphasis on safety data.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12085321/

Record 143

pmcid: PMC11744949

pmid: 39833706

doi: 10.1186/s12888-025-06493-0

title: A multidimensional assessment of adverse events associated with paliperidone palmitate: a real-world pharmacovigilance study using the FAERS and JADER databases.

authors

- Lou S
- Cui Z
- Ou Y
- Chen J
- Zhou L
- Zhao R
- Zhu C
- Wang L
- Wu Z
- Zou F

journal: BMC psychiatry

journalAbbr: BMC Psychiatry

year: 2025

pubdate: 2025 Jan 20

volume: 25

issue: 1

pages: 52

abstract: OBJECTIVE: Paliperidone palmitate is a second-generation antipsychotic that has undergone extensive investigation in clinical trials. However, real-world studies assessing its safety in large populations are lacking. As such, this study aimed to comprehensively evaluate real-world adverse drug events (ADEs) linked to paliperidone palmitate by employing data mining techniques on the U.S. Food and Drug Administration Adverse Event Reporting System (FAERS) database and the Japanese Adverse Drug Event Report (JADER) database. METHODS: The study retrieved ADE reports from the FAERS database covering the period from 2009 through the third quarter of 2024, and from the JADER database covering the period from 2013 through the second quarter of 2024. Utilizing disproportionality analyses such as the reporting odds ratios (ROR), proportional reporting ratios (PRR), Bayesian confidence propagation neural network (BCPNN), and multi-item Poisson shrinkage (MGPS), significant associations between ADEs and paliperidone palmitate were evaluated. RESULTS: A total of 27,672 ADE reports related to paliperidone palmitate were identified in FAERS, with 285 significantly disproportionate preferred terms (PTs) identified by all four algorithms. Paliperidone palmitate-associated ADEs encompassed 27 System Organ Classes (SOCs). The top three PTs with the highest reported cases were off-label use, drug ineffective, and hospitalization. Common ADEs included increased blood prolactin, galactorrhea, and schizophrenia, which was consistent with drug label. Noteworthy, unexpected signals not listed in the drug label were also identified, such as psychosexual disorders, prolactin-producing pituitary tumors, suicide attempt, and sudden death. The median onset time for all ADEs was 40 days. Furthermore, gender-based difference in risk signals was detected. Females are more likely to experience elevated blood prolactin and weight increase, whereas males are more prone to sexual dysfunction. Among the 1,065 ADE reports from the JADER database, we identified 51 positive signals, 35 of which overlapped with those found in FAERS, including schizophrenia, hyperprolactinemia, and erectile dysfunction. CONCLUSION: The study findings from two independent databases serve as crucial references for ensuring the safe of paliperidone palmitate. Additionally, the gender-specific monitoring references provided can enhance clinical

surveillance efforts and facilitate more effective risk identification.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11744949/

Record 144

pmcid: PMC11970347

pmid: 39853832

doi: 10.1111/hiv.13761

title: A programmatic assessment of dolutegravir adverse events and discontinuations in Uganda.

authors

- Odongpiny EAL
- Nicol M
- Katana E
- Owori J
- Buzibye A
- Sedan K
- Kesby M
- Holden M
- Owarwo N
- Meya D
- Castelnuovo B
- Sloan DJ
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journal: HIV medicine

journalAbbr: HIV Med

year: 2025

pubdate: 2025 Apr

volume: 26

issue: 4

pages: 592-605

abstract: INTRODUCTION: Dolutegravir is now extensively used in sub-Saharan Africa as a preferred component of antiretroviral therapy (ART). There is a paucity of large studies using routinely collected data from African people living with HIV on dolutegravir-based regimens to inform HIV programmes. We reviewed data in a large programme clinic of people living with HIV on dolutegravir to determine the real-world safety and tolerability of dolutegravir and to describe drivers of treatment discontinuation. METHODS: We carried out a retrospective dynamic cohort analysis of people living with HIV who started on or switched to dolutegravir-based ART at the Infectious Diseases Institute in Kampala, Uganda, between February 2017 and December 2020. Types of adverse events (AEs) were classified according to the Medical Dictionary for Regulatory Activities. Incident rates for AEs and treatment discontinuation were determined using Cox proportional hazard methods. RESULTS: Of 4529 people living with HIV started on or switched to dolutegravir, 2094 (45.9%) were female, and the median age was 49 years (interquartile range [IQR] 41-56). During 8907.93 person-years (PY) of follow-up, 1069 (23.6%; 95% confidence interval [CI] 22.4-24.8) people living with HIV had an AE, at a rate of 10.43 per 1000 PY (95% CI 9.77-11.14). Neuropsychiatric, gastrointestinal, and endocrine AEs were most common. The main AEs driving dolutegravir discontinuation were hyperglycaemia (140/356; 39.3%) and erectile dysfunction (19/356; 5.3%). Only 1.2% (4/356) of the dolutegravir discontinuations were because of neuropsychiatric AEs. Being female (adjusted hazard ratio [aHR] 1.35; 95% CI 1.02-1.78) and previous use of

stavudine (aHR 1.46; 95% CI 1.04-2.05) were the main predictors of neuropsychiatric AEs. Risk factors for hyperglycaemia included being overweight (aHR 1.66; 95% CI 1.11-2.47) or obese (aHR 1.84; 95% CI 1.12-3.05), having hypertension (aHR 1.92; 95% CI 1.35-2.73), having diabetes mellitus (aHR 12.6; 95% CI 8.34-19.1), and taking previous ART containing zidovudine (aHR 1.76; 95% CI 1.19-2.59) or stavudine (aHR 1.68; 95% CI 1.15-2.44). These risk factors for hyperglycaemia were also the main drivers of dolutegravir discontinuation. CONCLUSION: AEs were common in this African cohort, and dolutegravir discontinuation was driven by hyperglycaemia and erectile dysfunction. Previous use of older ART with known mitochondrial toxicity was associated with neuropsychiatric AEs and hyperglycaemia. African countries used these drugs for longer periods, and this may contribute to this risk.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11970347/

Record 145

pmcid: PMC12381642

pmid: 40881656

doi: 10.3389/fbioe.2025.1593368

title: A protocol for organoids from the gynecomastia patients.

authors

- Shang F
- Li Z
- Feng J
- Wang Q
- An M
- Zhao Z
- Liu B

journal: Frontiers in bioengineering and biotechnology

journalAbbr: Front Bioeng Biotechnol

year: 2025

pubdate: 2025

volume: 13

issue:

pages: 1593368

abstract: BACKGROUND: Gynecomastia, characterized by benign proliferation of male breast glandular tissue, is a prevalent condition with complex etiologies. However, the absence of effective in vitro models has hindered mechanistic investigations and therapeutic development. METHODS: In this study, we established and characterized organoids derived from the breast tissues of six male gynecomastia patients, including physiological, idiopathic, and hormone-related subtypes. Organoid fidelity was evaluated using hematoxylin and eosin (H&E) staining, immunohistochemistry (IHC), immunofluorescence (IF), and quantitative PCR (qPCR), targeting a panel of lineage-specific and proliferative markers. RESULTS: The organoids recapitulated key histological and molecular features of their corresponding source tissues, including epithelial architecture and expression of CK14, CK18, Ki67, and ERα. Marker expression was generally consistent between organoids and tissues at both the protein and transcriptional levels. Notably, ERα protein levels were reduced in organoids, while ESR1 mRNA expression remained stable, suggesting post-transcriptional regulation related to culture conditions. CONCLUSION: Our study presents a practical and reproducible protocol for generating gynecomastia-derived organoids and highlights their utility as a disease-relevant platform for future research in male breast pathology and hormone-related mechanisms.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12381642/

Record 146

pmcid: PMC12657040

pmid: 41311760

doi: 10.7759/cureus.95528

title: A Rare Case of Inflammatory Luminal B (HER2-Positive) Breast Cancer Misdiagnosed in a 30-Year-Old Woman: A Case Report.

authors

- Zablah JA
- Calderon JJ
- Burgos AN
- Gasca JA
- Montoya KA
- Orellana Banegas L
- Gracia M

journal: Cureus

journalAbbr: Cureus

year: 2025

pubdate: 2025 Oct

volume: 17

issue: 10

pages: e95528

abstract: A 30-year-old woman presented with a six-month history of a rapidly enlarging, painful mass in the left breast, accompanied by erythema and intermittent fever. Initial imaging suggested a benign lesion, with differential diagnoses including granulomatous mastitis and phyllodes tumor. She was treated empirically with antibiotics and corticosteroids, but her symptoms worsened. Follow-up imaging revealed further mass enlargement and persistent inflammatory changes. A core needle biopsy confirmed human epidermal growth factor receptor 2 (HER2)-positive, lymphocyte-poor invasive ductal carcinoma, Nottingham Grade III, with a high Ki-67 index and extensive necrosis. Staging studies showed no distant metastases. The patient is recommended to undergo neoadjuvant chemotherapy with HER2 targeted therapy, with plans for radical mastectomy and axillary lymph node dissection. This case underscores the diagnostic challenge of distinguishing inflammatory breast conditions. Inflammatory breast symptoms unresponsive to conservative treatment should prompt early biopsy and multidisciplinary evaluation to avoid delays in diagnosis, especially in young women presenting with atypical features.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12657040/

Record 147

pmcid: PMC11822275

pmid: 39958502

doi: 10.3348/jksr.2024.0054

title: A Rare Normal Variant with an Unusual Presentation on a Male Mammogram: A Case Report.

authors

- Tay VKL
- Tan SY
- Wong CY

- Leong LCH

journal: Journal of the Korean Society of Radiology

journalAbbr: J Korean Soc Radiol

year: 2025

pubdate: 2025 Jan

volume: 86

issue: 1

pages: 160-165

abstract: The sternalis muscle is a rare supernumerary muscle representing a normal anatomical variant in the anterior thoracic musculature. Due to wide variation in its morphology and relative unfamiliarity among radiologists, it has been implicated in the misdiagnosis of breast masses on mammography. A 23-year-old male with no significant medical history was referred to our institution for further management of painless bilateral breast enlargement since adolescence. Physical examination revealed breasts of slightly prominent size but there was no palpable breast lump. Mammography work-up found symmetrical, well-defined soft tissue masses projected over the posteromedial aspect of both breasts. Imaging findings were consistent with bilateral sternalis muscles, unusually hypertrophic in size due to intense upper body weight training by the patient. This case highlights the importance of recognizing the usual and unusual presentations of the sternalis muscles on mammography to avoid any unnecessary work-up.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11822275/

Record 148

pmcid: PMC12257316

pmid: 39344113

doi: 10.5534/wjmh.240029

title: A Review on Testosterone: Estradiol Ratio-Does It Matter, How Do You Measure It, and Can You Optimize It?

authors

- Swislocki ALM
- Eisenberg ML

journal: The world journal of men's health

journalAbbr: World J Mens Health

year: 2025

pubdate: 2025 Jul

volume: 43

issue: 3

pages: 453-464

abstract: There is a natural balance between the major sex steroids, testosterone and estradiol, controlled by gonadal secretion and peripheral conversion by aromatase. This balance is impacted by a variety of inborn and acquired conditions, and, more recently, by a growing use of exogenous testosterone therapy and off-label aromatase use under the guise of "men's health." We summarize reported testosterone:estradiol ratios, both naturally occurring and with pharmacologic manipulation and consider the ramifications of significant changes in these ratios. However, significant limitations exist in terms of steroid separation and measurement techniques, timing of samples, and lack of consistency from one assay to another, as well as definition of normative data. Limited data on the testosterone:estradiol ratio in men exists, particularly due to the scan data on concurrent estradiol values in men receiving testosterone therapy or aromatase inhibitors. Nonetheless, there seems to be

a range of apparently beneficial values of the testosterone: estradiol radio at between 10 and 30, calculated as: testosterone in ng/dL/estradiol in pg/mL. Higher values appear to be associated with improved spermatogenesis and reduced bone density while lower values are associated with thyroid dysfunction. While there is growing awareness of the significance of the testosterone:estradiol ratio, and a sense of a desired range, the optimal value has not yet been determined. Further work is needed to clarify the measurement strategies and clearly-defined outcome measures related to the testosterone:estradiol ratio.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12257316/

Record 149

pmcid: PMC12430505

pmid: 40942587

doi: 10.3390/ma18174161

title: Adaptive Sports Bra Design for Adolescents: A Flexible Fit Solution.

authors

- Kwan MY
- Zhong Z
- Yick KL
- Yip J
- Li NW
- Yu A
- Lo KW

journal: Materials (Basel, Switzerland)

journalAbbr: Materials (Basel)

year: 2025

pubdate: 2025 Sep 4

volume: 18

issue: 17

pages:

abstract: The development of adaptive and comfortable sports bras is essential for adolescents, who experience rapid changes in body morphology during growth. Traditional bras, often made with molded polyurethane bra pads, frequently fail to accommodate these variations, leading to discomfort and poor fit. This study investigates the design of a flexible-fit bra utilizing advanced knitting technology and bio-based materials, including organic cotton and renewable acetate, to enhance comfort and adaptability. The bra, crafted from bio-based yarns, offers stretchability, breathability, and fit, allowing it to adapt to various breast shapes and sizes. Such a bra design is particularly suitable for adolescents undergoing rapid growth. This study includes assessments of material properties and user feedback to evaluate the effectiveness of the design and identify areas for improvement. Positive results were reported from both material tests and subjective evaluations, confirming the effectiveness of the design. The seamless knitting minimizes irritation, while the inlay spacer fabric absorbs impact, and the pointelle structure improves moisture management. Adjustable components enhance adaptability and ensure a flexible fit. This study highlights the potential of knitted biomaterials for creating adaptive intimate apparel, offering a scalable solution for size-inclusive fashion.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12430505/

Record 150

pmcid: PMC12250665

pmid: 40644420

doi: 10.1371/journal.pone.0327842

title: Adverse event profile differences among long-acting gonadotropin-releasing hormone analogs: A real-world, pharmacovigilance study.

authors

- Chen Y
- Lu W
- Liao R
- Zhang X
- Chen W
- Wang J
- Feng H

journal: PloS one

journalAbbr: PLoS One

year: 2025

pubdate: 2025

volume: 20

issue: 7

pages: e0327842

abstract: BACKGROUND: Long-acting Gonadotropin-releasing hormone analogs(GnRHa), including leuprolide, goserelin, histrelin, buserelin, triptorelin, have been widely used for a variety of diseases including prostate cancer, breast cancer, endometriosis, uterine leiomyomas, and central precocious puberty (CPP). However, their real-world safety profile differences have not been adequately compared. OBJECTIVE: We aimed to investigate the adverse event (AE) profile differences of long-acting GnRHa reported by the US Food and Drug Administration Adverse Event Reporting System (FAERS). METHODS: All indications were searched long-acting GnRHa, as primary suspect drugs, from FAERS data (January 2004 to September 2023). We performed disproportionality analyses by reporting odds ratios (ROR) and conducted univariate and multivariate logistical regression analyses to determine the odds ratio (OR) of serious AEs associated with long-acting GnRHa under different exposure factors. RESULTS: Reproductive system and breast disorders accounted for the greatest proportion of adverse events among the five long-acting GnRHa formulations analyzed. Both buserelin and histrelin showed distinct adverse effect profiles, with buserelin demonstrating a higher incidence of gastrointestinal disorders and histrelin showing a greater propensity for psychiatric disorders. Logistic regression analysis revealed these five medications carried an elevated risk of significant medical events, and this risk was notably lower in pediatric patients (<18 years) compared to adult populations (≥18 years). CONCLUSIONS: Significant disparities exist between the adverse event profiles of long-acting GnRHa. The identification of high-risk factors and the enhancement of AEs monitoring are crucial during clinical application.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12250665/

Record 151

pmcid: PMC12011079

pmid: 40259991

doi: 10.1093/sexmed/qfaf023

title: AI-assisted transcription of YouTube videos on penile enlargement: analysis of their text quality and readability.

authors

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journal: Sexual medicine

journalAbbr: Sex Med

year: 2025

pubdate: 2025 Apr

volume: 13

issue: 2

pages: qfaf023

abstract: BACKGROUND: Patients dealing with sensitive issues like penile enlargement (PE) might benefit from YouTube videos. Therefore, it is essential that the textual content of these videos is clear, trustworthy, and of high quality. AIM: Are the AI-assisted acquired texts' qualities and comprehensibilities of YouTube videos about PE enough and suitable for the patients? METHODS: On October 25, 2024, Google Trends analysis identified the 25 most searched phrases for "Penile enlargement." Non-related terms were excluded, and the relevant keywords were searched on YouTube. Only content about PE included; excluding duplicates, non-English videos, YouTube shorts, those under 30 seconds, silent, and music-only videos. Videos were transcribed using Whisper AI, and their quality was assessed by M.F.Ş, E.C.T., and Ç.D. using the GQS (global quality scale) and DISCERN, the readability was evaluated via Flesch-Kincaid (FKGL and FKRE) measures. High assessor agreement was noted (Pearson r = 0.912). Videos were categorized by uploader, and metrics such as views, likes, comments, and duration were recorded. The Chi-square test was used for categorical variable comparisons; the Kruskal-Wallis H-Test was applied when normality and homoscedasticity were not met, with Bonferroni post hoc correction for multiple comparisons. OUTCOMES: The mean DISCERN and GQS scores were 51.23 ± 13.1 and 3.32 ± 0.9, respectively. FKRE and FKGL scores were 73.12 ± 11.7 and 5.85 ± 2.1. Physicians (n = 67) produced the most videos, while academic institutions (n = 2) produced the least. No significant differences in text quality were found between groups (P = 0.067 and P = 0.051). Health-related websites exhibited lower FKRE compared to non-healthcare videos (P = 0.002), with a significant difference in FKGL as well (P = 0.019). RESULTS: The video exhibited a high level of readability (indicating comprehensibility for almost a 6th-grade student). Text quality, view and like count of the videos uploaded by academic institutions was the highest. CLINICAL IMPLICATIONS: In PE, YouTube video's health information needs to be better quality and more trustworthy, according to our research. The language used in videos should be easier to understand. STRENGTHS AND LIMITATIONS: This study is the first scientific analysis of YouTube video transcripts on PE using AI, focusing specifically on English content, which limits its applicability to non-English speakers and other platforms. Exclusions of silent and shorter videos may result in the omission of valuable information. CONCLUSION: The need for better quality and trustworthiness in health-related YouTube information, especially for PE is essential. Content makers should stress clear, accessible language and minimize disinformation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12011079/

Record 152

pmcid: PMC12643153

pmid: 41293137

doi: 10.4103/ija.ija_1082_25

title: All India Difficult Airway Association 2025 guidelines for the management of unanticipated difficult airway in obstetrics under general anaesthesia.

authors

- Ramkumar V
- Jain K
- Shah AP
- Shetty SR
- Bhat MG
- Parameswari A
- Ahmed SM
- Grewal A
- Myatra SN
- Garg R
- Doctor JR
- Divatia JV
- Das S
- Kundra P

journal: Indian journal of anaesthesia

journalAbbr: Indian J Anaesth

year: 2025

pubdate: 2025 Nov

volume: 69

issue: 11

pages: 1142-1166

abstract: Central neuraxial anaesthesia is the preferred approach for caesarean deliveries leading to less frequent use of general anaesthesia. The 2025 guidelines for managing general anaesthesia in obstetrics recommend rapid sequence induction and intubation for parturients requiring general anaesthesia for caesarean delivery. If the initial intubation attempt fails, the team should "Call for help for difficult airway" (Code D) and prioritise oxygenation (SpO 2 > 95%). Tracheal intubation may be attempted by a more experienced operator after re-optimising position and considering alternate airway devices. The primary anaesthesia team can opt for a 2 nd generation supraglottic airway (SGA) device as a rescue device or continue the anaesthetic using a face mask (FM). Maintaining oxygenation is critical. If satisfactory oxygenation is achieved with 2 nd generation SGA or FM, the decision to continue will depend upon maternal and fetal condition. If satisfactory ventilation cannot be maintained with 2 nd generation SGA or FM following failed tracheal intubation, the team declares "complete ventilation failure" and proceeds to surgical cricothyroidotomy. Should maternal cardiac arrest occur, proceed to "resuscitative hysterotomy" to improve chances of saving mother and baby. Post-resuscitation care should continue in the ICU to return the parturient to normalcy. Parturients may have a physiologically difficult airway in addition to anatomical reasons for airway difficulty. Specific steps to deal with associated pathophysiological changes constitute an important aspect of airway management in obstetrics. If a difficult airway is encountered, an "Airway Alert Card" must be given to the parturient and her responsible relatives to avoid similar life-threatening situations in future.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12643153/

Record 153

pmcid: PMC12493217

pmid: 41050522

doi: 10.1016/j.eucr.2025.103211

title: An unusual path to diagnosis: Sertoli cell neoplasia revealing Peutz-Jeghers syndrome in an 8-year-old

boy.

authors

- Almusalam R
- Janahi S
- Basem M
- Isa H
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journal: Urology case reports

journalAbbr: Urol Case Rep

year: 2025

pubdate: 2025 Nov

volume: 63

issue:

pages: 103211

abstract: Sertoli cell tumors (SCTs) are a rare occurrence, accounting for 0.4 %-1.5 % of all testicular masses. They are characterized as classic, large cell calcifying, or sclerosing Sertoli cell tumors depending on their clinical characteristics. These tumors can be associated with multiple genetic disorders, one of which is Peutz-Jeghers syndrome (PJS). PJS is a result of the mutation of STK11/LKB1 gene, manifesting as oral hyperpigmentation, benign colonic polyps, gastrointestinal and non-gastrointestinal tumors. Here we describe a case of intratubular large cell hyalinizing Sertoli cell tumor, a particularly rare variant of SCTs, with the detailed steps in diagnosis and management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12493217/

Record 154

pmcid: PMC12443299

pmid: 40961031

doi: 10.1371/journal.pone.0330659

title: Analyzing real-world adverse events of spironolactone with the FAERS database.

authors

- Lin G
- Chen R
- Wen C
- Li Z
- Yan X
- Li L

journal: PloS one

journalAbbr: PLoS One

year: 2025

pubdate: 2025

volume: 20

issue: 9

pages: e0330659

abstract: BACKGROUND: Spironolactone, a potassium-sparing diuretic, is commonly prescribed for conditions such as heart failure, hypertension, and hyperaldosteronism. This study aims to explore and analyze the safety profile of spironolactone by examining adverse event reports from the FDA Adverse Event Reporting System (FAERS) database. METHODS: This study conducted a retrospective pharmacovigilance analysis using FAERS data (2004 Q1-2024 Q3). Adverse drug events (ADEs) related to spironolactone were identified and categorized by system organ class and specific adverse events. Statistical methods such as Proportional Reporting Ratio (PRR), Reporting Odds Ratio (ROR), Bayesian Confidence Propagation Neural Network (BCPNN), and Empirical Bayesian Geometric Mean (EBGM) were employed to detect potential safety signals. RESULTS: A total of 8,566 ADE reports were associated with spironolactone, with 2409 preferred terms and 25 system organ classes showing significant disproportionality. Notable rare ADEs identified included endometriosis male (n = 7; ROR 13615.84), 5-alpha-reductase deficiency (n = 5; ROR 1620.81), bulbospinal muscular atrophy congenital (n = 6; ROR 402.42) and double-hit lymphoma (n = 5; ROR 243.12). CONCLUSION: While most findings were consistent with spironolactone's known effects, new signals, including a potential link to male endometriosis in high-risk groups, were identified. Further research is needed to confirm these findings and improve long-term safety assessment and clinical management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12443299/

Record 155

pmcid: PMC12683080

pmid: 41367474

doi: 10.7759/cureus.96346

title: Angiomatosis of the Breast: Unveiling a Rare and Unusual Vascular Puzzle.

authors

- Aishwarya Teja A
- Damodaran A
- Pitambra A
- Nigam JS
- Kumar S

journal: Cureus

journalAbbr: Cureus

year: 2025

pubdate: 2025 Nov

volume: 17

issue: 11

pages: e96346

abstract: Angiomatosis of the breast is a very rare benign vascular lesion that often resembles malignant vascular tumors both clinically and radiologically. Accurate recognition is crucial to avoid misdiagnosis and unnecessary interventions. We report the case of a 35-year-old female who presented with a 20-year history of recurrent, painless enlargement of the left breast. A nipple-sparing mastectomy was performed, and the specimen, measuring 29.5 × 17.5 × 2.5 cm, revealed a diffuse spongy lesion with multiple cystic spaces. Microscopically, the lesion was composed of anastomosing vascular channels of variable caliber surrounding but not invading breast lobules. Immunohistochemistry demonstrated strong nuclear positivity for FLI1 and cytoplasmic positivity for CD31 and CD34, with weak or absent CD34 expression in some vascular spaces. The Ki-67 proliferation index was <2%. These findings suggest the diagnosis of breast angiomatosis. The postoperative recovery was smooth, and no recurrence was observed after four months of follow-up. These cases must be distinguished from low-grade angiosarcoma, which shares overlapping histological features. Features such as the absence of cytologic atypia, a low proliferative index, and supportive immunohistochemical findings help confirm the diagnosis. Complete surgical excision with clear margins is essential, as incomplete

excision carries a risk of local recurrence. This report highlights the importance of thorough histopathological and immunohistochemical evaluation in differentiating angiomatosis from malignant vascular tumors of the breast.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12683080/

Record 156

pmcid: PMC12213018

pmid: 40605923

doi: 10.1055/s-0045-1801866

title: Aquafilling Filler for Buttock Augmentation Cause Severe Long-Term Complications: A Case Report.

authors

- Kristiansen CS
- Lang CL
- Haerskjold A
- Zachariae C
- Klit A

journal: Indian journal of plastic surgery : official publication of the Association of Plastic Surgeons of India

journalAbbr: Indian J Plast Surg

year: 2025

pubdate: 2025 Jun

volume: 58

issue: 3

pages: 222-225

abstract: Filler injections for buttock augmentation are becoming more popular as a cosmetic procedure, which leads to an expected increase in the number of complications emphasizing the importance of reporting cases with both well-known, severe, and previously undiscovered complications, as well as their possible treatment strategies. We present an 18-year-old woman who suffered severe long-term complications following Aquafilling injections for buttock augmentation, including filler migration, infection leading to septic shock, and nonparathyroid hypercalcemia, which has the potential to cause renal insufficiency. To date, we have not found any reports describing the association between nonparathyroid hypercalcemia and Aquafilling. Additionally, we outline a treatment regimen involving a minimally invasive approach, which includes daily irrigation, manual compression, and passive evacuation. At follow-up, the patient returned to her everyday life with no lasting sequelae, except for a solid mass medially on the right thigh.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12213018/

Record 157

pmcid: PMC12579050

pmid: 41180436

doi: 10.1007/s13224-025-02165-x

title: Assessment and Establishment of Determinants of Sexual Activity in Pregnancy in Primigravidae and Multigravidae.

authors

- Gandhi AA
- Samant PY

journal: Journal of obstetrics and gynaecology of India

journalAbbr: J Obstet Gynaecol India

year: 2025

pubdate: 2025 Oct

volume: 75

issue: 5

pages: 410-414

abstract: BACKGROUND: Pregnancy is a special period in a woman's life that involves physical and mental changes. This study aimed at evaluating woman's sexual experience in pregnancy. METHODS: Cross sectional study design. We collected data pertaining to pregnant women's sexual behaviour via a questionnaire from 171 healthy pregnant women. Responses were summarised and comparisons were made between trimesters of pregnancy and primigravidae and multigravidae. RESULTS: We studied various aspects such as marriage related information, factors affecting sexual activity, frequency of sexual activity across trimesters, causes for abstinence of sexual activity in primigravidae and multigravidae. The majority of women perceived an overall decline in sexual desire progressively increasing in each trimester. None of the patients reported an increase in coital frequency. We determined that in 62 cases, the source of information regarding sexual activity during pregnancy was their partners. CONCLUSION: The authors encourage the provision of sexuality education to newlyweds and the resolution of sex-related concerns during pregnancy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12579050/

Record 158

pmcid: PMC12784444

pmid: 41523426

doi: 10.7759/cureus.98881

title: Atypical Chest Wall Cystic Hygroma in a Toddler: A Case Report and Comprehensive Review.

authors

- Askar AS
- Mohamed SF
- Taha A
- Obeydan M
- Nassan S

journal: Cureus

journalAbbr: Cureus

year: 2025

pubdate: 2025 Dec

volume: 17

issue: 12

pages: e98881

abstract: Lymphangiomas, also known as cystic hygromas, are rare benign lymphatic malformations that typically occur in the head and neck region. Breast or chest wall involvement in pediatric patients is exceptionally rare, with only a few cases reported in the literature. We report a case of a three-year-old boy who presented with a progressively enlarging right breast mass. Unsupervised topical creams and massage led to further enlargement. Laboratory investigations showed mild elevation in prolactin levels, which normalized after surgery. Ultrasonography demonstrated multiple cystic lesions, the largest of which was oval and measured 26 mm in diameter, along with an enlarged right axillary lymph node. Computed tomography (CT) scan showed a

well-defined oval mass (51 × 26 mm) in the right chest wall. The patient underwent surgical excision, and a complete histopathological examination was performed. Histopathological examination revealed multiple lymphatic spaces lined by flattened endothelial cells forming a tumorous mass, with both cavernous and capillary-type lumens. The diagnosis of lymphangioma was confirmed. No mammary tissue was detected, and cancer was excluded. Subsequent evaluations over one week indicated full recovery with normalized prolactin levels. This case underscores the necessity of including lymphangioma in the differential diagnosis of young chest wall tumors. Although this condition is uncommon, early detection and appropriate surgical intervention result in favorable outcomes with a low chance of recurrence.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12784444/

Record 159

pmcid: PMC13096872

pmid: 40103466

doi: 10.2174/0115734056347327250117073638

title: Background Parenchymal Enhancement in Breast MRI Correlates with Molecular Subtypes of Breast Cancer.

authors

- Liu H
- Chen X
- Wang Y
- Yang X
- Chen Y

journal: Current medical imaging

journalAbbr: Curr Med Imaging

year: 2025

pubdate: 2025

volume: 21

issue:

pages: e15734056347327

abstract: PURPOSE: MRI could be considered as a non-destructive disease diagnosis procedure, this procedure does not allow directly molecular types of cancer. Herein, we aimed to evaluate the correlation of breast MRI background parenchymal enhancement (BPE) and fibroglandular tissue (FGT) with the molecular subtypes and immunohistochemical markers of breast cancer. METHODS: This was a single-cross-sectional retrospective study.Fifty-six patients diagnosed with unilateral breast cancer who underwent breast MRI scans before needle biopsy or surgery were selected. The relationship between qualitative and quantitative BPE/FGT ratios and the expression of breast cancer molecular subtypes and immunohistochemical markers were evaluated in patients with breast cancer. RESULTS: Quantitative BPE (BPE%) of luminal A and luminal B was significantly lower than that of triple-negative breast cancer. There was no significant difference in the qualitative BPE/FGT between the different breast cancer subtypes. The quantitative BPE (BPE%) of estrogen receptor (ER)- negative tumors was higher than that of the ER-positive tumors, and the expression of FGT%, BPE%, and other immunohistochemical markers (human epidermal growth factor receptor-2(HER-2), progesterone receptor (PR), and Ki-67) were not significantly different. The proportion of high BPE distribution in HER-2 positive tumors was higher than that in the HER-2 negative group; however, there was no significant difference in the expression of qualitative BPE/FGT and other immunohistochemical markers (ER, PR, and Ki-67). CONCLUSION: There were significant differences in the levels of BPE among the different molecular subtypes. Therefore, BPE may be a potential imaging biomarker for the diagnosis of the molecular subtypes of breast cancer.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13096872/

Record 160

pmcid: PMC12535809

pmid: 41116907

doi: 10.1155/crpe/5815092

title: Bifocal β-hCG-secreting CNS Germinoma in a 13-Year-Old Boy: Clinical-Biochemical Pubertal Discordance and Long-Term Outcome.

authors

- Nicolescu CR
- Doulet ST
- Bazus L
- Stephan JL

journal: Case reports in pediatrics

journalAbbr: Case Rep Pediatr

year: 2025

pubdate: 2025

volume: 2025

issue:

pages: 5815092

abstract: BACKGROUND: Intracranial germ cell tumors (GCTs) are rare in the pediatric population. They are classified as germinoma and nongerminomatous and may secrete specific proteins such as β subunit of human chorionic gonadotropin (β-hCG) and alpha-fetoprotein (AFP). When secreting β -hCG, they may induce gonadotropin-independent puberty (GIP), a clinical diagnostic clue that can precede neuroimaging findings. CASE: A 13-year-old boy presented with a first generalized tonic-clonic seizure after six months of headaches, vomiting, polyuria, and polydipsia. Examination showed pubertal penile length with peripubertal testes. Laboratory assessment revealed panhypopituitarism with suppressed gonadotropins and elevated testosterone. Brain magnetic resonance imaging (MRI) demonstrated bifocal lesions (pineal and suprasellar) with obstructive hydrocephalus. Cerebrospinal fluid (CSF) sample obtained via temporary external ventricular drain (EVD) showed a normal cytology, confirming a β -hCG-secreting germinoma. The patient achieved remission after chemotherapy, neurosurgical resection, and cranial radiotherapy. He developed a posterior medulla relapse successfully salvaged with gemcitabine-paclitaxel-oxaliplatin, high-dose etoposide-thiotepa with autologous stem-cell transplantation, and craniospinal irradiation. CONCLUSION: Discordant clinical and biochemical puberty (penile enlargement with small testes and high testosterone with suppressed gonadotropins) should prompt β -hCG testing and targeted neuroimaging for suspected central nervous system GCTs. When raised intracranial pressure precludes lumbar puncture, CSF sampling via EVD is a safe alternative. Coordinated oncologic-endocrine care supports durable disease control with tailored long-term hormonal follow-up.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12535809/

Record 161

pmcid: PMC11952889

pmid: 40162139

doi: 10.1093/omcr/omaf014

title: Bilateral breast lymphoma in lactating woman: rare and particularly aggressive entity.

authors

- Lahlimi FE
- Abakarim O

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journal: Oxford medical case reports

journalAbbr: Oxf Med Case Reports

year: 2025

pubdate: 2025 Apr

volume: 2025

issue: 4

pages: omaf014

abstract: Non-Hodgkin's lymphoma (NHL) is seldom encountered in breast. Primary malignant lymphomas of the breast in pregnant or lactating women are more uncommon with massive bilateral breast enlargement rapidly followed by widespread dissemination to several organs. Its diagnosis is often delayed, its treatment still be defined, and its prognosis is particularly poor. We report the case of a 20-year-old Moroccan woman with bilateral primary diffuse B-cell lymphoma of the breast, treated with chemotherapy without a favorable response, progressed to deterioration then death. This case highlights the management difficulties of this group of very high-risk lymphomas.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11952889/

Record 162

pmcid: PMC12740280

pmid: 40437888

doi: 10.4274/ejbh.galenos.2025.2025-4-4

title: Bilateral Gestational Gigantomastia Complicating Pregnancy: A Challenging Case Refractory to Conservative Management.

authors

- Pravanan S
- Grero L
- Wijerathna W
- Ranaweera K
- Senavirathna J
- Sanjeewa SHR
- Wijesinghe K

journal: European journal of breast health

journalAbbr: Eur J Breast Health

year: 2025

pubdate: 2025 May 29

volume: 22

issue: 1

pages: 98-101

abstract: Gestational gigantomastia (GG) is a rare condition characterized by excessive and rapid breast enlargement during pregnancy, resulting in significant physical discomfort, functional limitations, and significant psychological impact. We present a case of a 33-year-old multiparous woman in her third pregnancy, who developed severe bilateral GG by 16 weeks of gestation. Despite initial conservative management, including

analgesia and pharmacological (bromocriptine) therapy, the condition worsened causing functional impairment and recurrent mastitis requiring repeated hospital admissions. The pregnancy was electively induced due to physical limitations at 35 weeks of gestation; however, the labour was complicated by obstruction, necessitating an emergency Cesarean section. Postpartum the patient developed severe lactational mastitis complicated by sepsis necessitating intensive care unit admission. After recovery and cessation of breastfeeding, she elected to undergo Wise-pattern bilateral reduction mammoplasty with free nipple-areolar complex grafting four months into her postpartum period. The procedure provided substantial functional relief and a favorable esthetic outcome. This case highlights the potential complexity of managing GG and the need for individualized care. Although conservative treatments may offer temporary relief, surgical intervention is often necessary in severe cases.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12740280/

Record 163

pmcid: PMC12223670

pmid: 40631651

doi: 10.5765/jkacap.250006

title: Body Dysmorphic Disorder and Risperidone Abuse: Exploiting Hyperprolactinemia.

authors

- Bulut ÖF

journal: Journal of the Korean Academy of Child and Adolescent Psychiatry

journalAbbr: J Korean Acad Child Adolesc Psychiatry

year: 2025

pubdate: 2025 Jul 1

volume: 36

issue: 3

pages: 182-184

abstract: This case report describes a 16-year-old female with body dysmorphic disorder (BDD), tics, and social withdrawal who intentionally abused risperidone to exploit its hyperprolactinemia-induced breast enlargement, perceiving it as beneficial to her body image. Initial treatment with sertraline improved social withdrawal; however, adjunctive risperidone (0.5 mg/day) reduced tics and BDD symptoms. Elevated prolactin levels (44.85 ng/mL) triggered breast enlargement, galactorrhea, and secondary amenorrhea. The patient continued risperidone despite medical advice to discontinue it. Switching to aripiprazole normalized prolactin levels (9.13 ng/mL) and resolved the side effects, with sustained symptom control at 3-month follow-up. This case highlights the risk of antipsychotic side effects reinforcing BDD pathology and adolescents' vulnerability to medication abuse when somatic changes align with dysmorphic goals. To the best of our knowledge, this is the first report of deliberate risperidone abuse for cosmetic hyperprolactinemia in BDD. Clinicians must monitor prolactin levels, prioritize prolactin-sparing agents (e.g., aripiprazole), and consider how adverse effects may perpetuate body image disturbances in adolescents.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12223670/

Record 164

pmcid: PMC12281992

pmid: 40691558

doi: 10.1186/s12905-025-03908-8

title: Breast carcinoma in a patient with neurofibromatosis type 1 and huge plexiform neurofibroma of the contralateral breast: a case report.

authors

- Kouli A
- Alayoubi A
- Alali M
- Saifo M

journal: BMC women's health

journalAbbr: BMC Womens Health

year: 2025

pubdate: 2025 Jul 21

volume: 25

issue: 1

pages: 360

abstract: BACKGROUND: Neurofibromatosis type 1 (NF1) is a genetic disorder associated with an increased risk of various cancers, including breast cancer. This report presents a case of a patient with NF1 who had a huge plexiform neurofibroma of the right breast and developed invasive carcinoma in the left breast. CASE PRESENTATION: A woman with a known history of NF1, enlarged right breast, and pectus carinatum presented with locally advanced breast cancer of the left breast. The patient underwent four cycles of neoadjuvant chemotherapy with doxorubicin and cyclophosphamide, followed by a left modified radical mastectomy and axillary lymph node dissection. Postoperative pathology showed a complete pathological response. Subsequently, the patient received four cycles of adjuvant paclitaxel followed by endocrine therapy with anastrozole which was replaced by tamoxifen due to bone and muscle pain. At one year follow-up, the patient remains free of disease. The patient was referred to a plastic surgeon for resection of the enlarged right breast. CONCLUSION: It is known that women with NF1 have a higher risk of developing breast cancer, as demonstrated in our case. These patients could benefit from an early start of breast cancer screening, which could lead to an early diagnosis of early-stage tumors and a better prognosis. Additionally, enhanced access to healthcare centers and intensive surveillance could contribute significantly to better outcomes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12281992/

Record 165

pmcid: PMC12150844

pmid: 40496032

doi: 10.2478/jcas-2025-0005

title: Breast Hamartoma with Unusual Imaging Features.

authors

- Gul P
- James J
- Hajaj M
- AlNajjar D

journal: Journal of cancer & allied specialties

journalAbbr: J Cancer Allied Spec

year: 2025

pubdate: 2025 Jan

volume: 11

issue: 1

pages: 29-33

abstract: INTRODUCTION: Hamartomas are benign masses seen in different organs including breasts. These are well-defined masses with a pseudocapsule and variable tissues from the surrounding organ.It is an uncommonly reported benign breast lesion, the cause and pathogenesis of its development is still not fully understood. The tumour exhibits a female predominance with a mean age of 45. CASE DESCRIPTION: We present two cases of breast hamartoma in 51 and 61 years old female who presented to us through screening and symptomatic clinics.Both cases had imaging findings typical for diagnosis of hamartoma however there was a distortion within the hamartoma which raised possibility of malignancy and prompted a core biopsy and MDT discussion. PRACTICAL IMPLICATION: Many hamartomas have typical benign imaging appearances and require no further investigation. Biopsy is needed in atypical looking hamartomas to exclude a breast carcinoma, which although rare may develop within a hamartoma. Multidisciplinary team discussion is crucial in the management of atypical cases to ensure radiological-pathological concordance.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12150844/

Record 166

pmcid: PMC12180105

pmid: 40421493

doi: 10.4274/ejbh.galenos.2025.2025-4-3

title: Breast Lymphedema Secondary to Lymph Node Tuberculosis: Case Report.

authors

- Vaquero-Ramiro E
- Puentes-Gutiérrez AB
- Millán-Casas L
- García-Bascones M

journal: European journal of breast health

journalAbbr: Eur J Breast Health

year: 2025

pubdate: 2025 Jun 20

volume: 21

issue: 3

pages: 274-276

abstract: Breast lymphedema is a common but underdiagnosed condition that affects the quality of life of patients. It may be caused by any pathology that disrupts lymphatic drainage in the breast. We present the case of a woman with axillary lymph node tuberculous infection with breast edema, clinically and radiographically indistinguishable from tuberculous mastitis. After six months of comprehensive antituberculosis pharmacological treatment, the persistence of breast edema required repeating diagnostic tests searching for malignancy, all of which were negative. Rehabilitation treatment with complex physical therapy improved the patient's clinical and symptomatic condition. Clinical suspicion of secondary lymphedema is crucial to avoid unnecessary diagnostic procedures and ensure adequate and timely treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12180105/

Record 167

pmcid: PMC12068584

pmid: 40354418

doi: 10.1371/journal.pone.0321205

title: Breast tenderness and swelling experiences related to menstrual cycles and ovulation in healthy

premenopausal women: Secondary analysis of the 1-year "Prospective Ovulation Cohort".

authors

- Wood M
- Shirin S
- Goshtasebi A
- Prior JC

journal: PloS one

journalAbbr: PLoS One

year: 2025

pubdate: 2025

volume: 20

issue: 5

pages: e0321205

abstract: Breast tenderness and swelling are associated with premenstrual symptoms but are not well described in healthy women. In this 1-year prospective observational study, we examined daily breast tenderness and swelling to determine whether differences existed between normally ovulatory and ovulatory disturbed (short luteal phase and anovulatory) cycles in a cohort of community dwelling, non-smoking, healthy premenopausal women. Enrolment required two consecutive normal-length and normally ovulatory cycles by Quantitative Basal Temperature© analysis. Women (n = 53) ages 20-41 recorded their daily breast experiences in the Menstrual Cycle Diary© across an average of 13.6 cycles. In all 720 cycles, the median breast tenderness was 1.4 (on a 0-4 scale, range 0.0-3.0), in cycles with a mean length of 28.1 days (95% CI 27.5-28.8). Comparison of breast tenderness and breast size (changes from usual) parameters between all normally ovulatory cycles and all ovulatory disturbed cycles in the whole cohort showed significantly higher levels in normally ovulatory (luteal length ≥10 days) in both Breast Tenderness Score [intensity X duration in days; 6.0 (range 1.0-14.0) vs. 3.0 (0.0-11.0) (P=.005)] and breast size [4.0 (2.0-4.0) vs. 4.0 (0.0-4.0) (P=.034]). However, within-woman in the forty-seven women with both normally ovulatory and ovulatory disturbed cycles, breast tenderness (intensity, duration, and Breast Tenderness Score), did not differ between normally ovulatory cycles and cycles with ovulatory disturbances. This study also demonstrated that in all ovulatory cycles, the timing of breast tenderness increased in parallel with breast swelling; the maximum for both was in the late luteal phase.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12068584/

Record 168

pmcid: PMC12597514

pmid: 40623009

doi: 10.1093/jnci/djaf177

title: Breastfeeding after breast cancer in young BRCA carriers.

authors

- Blondeaux E
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- Cortesi L
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- Yerushalmi R
- Rodriguez-Wallberg KA
- Renaud T
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- Wong SM
- Pogoda K
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- Puglisi F
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journal: Journal of the National Cancer Institute

journalAbbr: J Natl Cancer Inst

year: 2025

pubdate: 2025 Nov 1

volume: 117

issue: 11

pages: 2229-2239

abstract: BACKGROUND: We investigated safety of breastfeeding after breast cancer in patients carrying

germline BRCA pathogenic or likely pathogenic variants. METHODS: This was an international, multicenter, hospital-based, retrospective cohort study including BRCA carriers diagnosed with stage I-III invasive breast cancer at age 40 years or younger between January 2000 and December 2020 (NCT03673306). Locoregional recurrences and/or contralateral breast cancers, disease-free survival (DFS), and overall survival (OS) were compared between patients who breastfed after delivery and those who did not. RESULTS: Among 4732 patients included from 78 centers worldwide, 659 had a pregnancy after breast cancer diagnosis, of whom 474 delivered a child. After excluding patients with uptake of bilateral risk-reducing mastectomy prior to delivery (n = 225) or unknown breastfeeding status (n = 71), 110 (61.8%) breastfed (median duration 5 months) and 68 (38.2%) did not breastfeed. Compared to patients in the no breastfeeding group, those who breastfed were more frequently nulliparous at breast cancer diagnosis (61.8% vs 45.6%) and did not report prior smoking habit (71.8% vs 57.4%). After a median follow-up of 7.0 years following delivery, 7-year cumulative incidence of locoregional recurrences and/or contralateral breast cancers was 29% in the breastfeeding group and 36% in the no breastfeeding group (adjusted subdistribution hazard ratio [HR] = 1.08, 95% CI = 0.57 to 2.06). No difference in DFS (adjusted hazard ratio [aHR] = 0.83, 95% CI = 0.49 to 1.41) nor in OS (aHR = 1.32, 95% CI = 0.31 to 5.66) was observed. CONCLUSIONS: Breastfeeding did not appear to be associated with a higher risk of developing locoregional recurrences or contralateral breast cancers, emphasizing the possibility of achieving a balance between maternal and infant needs without compromising oncological safety.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12597514/

Record 169

pmcid: PMC12103448

pmid: 40420883

doi: 10.1007/s12070-025-05487-4

title: Can Clinical/ Endocrinal Surrogates Predict Specific Hormone Therapy in Nasopharyngeal Angiofibroma? Current Evidence.

authors

- Pathak P
- Mishra A
- Mishra SC

journal: Indian journal of otolaryngology and head and neck surgery : official publication of the Association of Otolaryngologists of India

journalAbbr: Indian J Otolaryngol Head Neck Surg

year: 2025

pubdate: 2025 Jun

volume: 77

issue: 6

pages: 2283-2292

abstract: UNLABELLED: Objective: In an attempt to rationalize Hormone therapy (HT) in juvenile nasopharyngeal angiofibroma (JNA) this study intends to investigate the pituitary axis and clinical profile to further establish dominant hormonal-environment based upon secondary sexual characteristics along with other novel possibilities for HT. Design: Prospective observational study. Setting: Tertiary health care centre. Participants: Thirty-three patients were analysed in terms of age, secondary sexual characteristics (SSC), staging, preoperative serum-hormone profile and weight/ volume of excised tumour. Main outcome measures: Serum levels of prolactin, testosterone, oestrogens, progesterone, LH and secondary sexual characteristics. Results: Significant correlation was seen between LH/ testosterone/ estrogen on one hand while LH/ GH/ progesterone on other. LH/ testosterone correlated with age, tumour weight, & almost all SSC. Oestrogen correlated with age, & all SSC (except facial hair/ laryngeal prominence/ deepening of voice). Accordingly, 2 sets of SSC (clinical surrogate markers) defined estrogenic or androgenic predominance in JNA. Conclusions: The clinical surrogates may indirectly reflect corresponding receptor activity (rather than isolated hormone level)

and provide a better rationale for selecting a particular HT. SUPPLEMENTARY INFORMATION: The online version contains supplementary material available at 10.1007/s12070-025-05487-4.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12103448/

Record 170

pmcid: PMC12568417

pmid: 41170446

doi: 10.3389/fonc.2025.1652446

title: Case report: Adolescent breast hypertrophy with a tendency toward phyllodes tumor formation: first reported case in China.

authors

- Liao R
- Xiao Y
- Chang Y
- Zhang Z
- Zhang Y

journal: Frontiers in oncology

journalAbbr: Front Oncol

year: 2025

pubdate: 2025

volume: 15

issue:

pages: 1652446

abstract: Adolescent breast hypertrophy is a rare and persistent condition that affects peripubertal females both physiologically and psychologically. We present a case of a 14-year-old patient with bilateral giant breast enlargement who underwent a successful reduction mammoplasty of 16.5% Body Weight. The right breast exhibited focal interstitial proliferation myofibroblasts, suggesting a potential risk for phyllodes tumor development. This finding represents a novel histopathological association, unreported in the literature and first documented in China.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12568417/

Record 171

pmcid: PMC11926138

pmid: 40125530

doi: 10.3389/fgene.2025.1523228

title: Case Report: Area of focus clinical presentation and KMT2D gene mutation at the c.15535C>T site in a case of Kabuki syndrome.

authors

- Li W
- Lin M
- Dao J
- Shi L
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journal: Frontiers in genetics

journalAbbr: Front Genet

year: 2025

pubdate: 2025

volume: 16

issue:

pages: 1523228

abstract: BACKGROUND: Kabuki syndrome (KS) is a rare autosomal dominant genetic disorder. The full understanding of KS remains elusive due to the heterogeneity of gene mutations, clinical phenotypes, and the associations and mechanisms linking genotypes to phenotypes. This study reports on a 16-year-old male patient diagnosed with type I Kabuki syndrome following the identification of a de novo mutation, c.15535C>T (p.Arg5179Cys), in the KMT2D gene. CASE REPORT: A 16-year-old male presented with bilateral breast enlargement persisting for over 1 month. Historically, the patient exhibited intellectual disability. Both parents are healthy with no similar family history. The patient's father had a history of heroin use for 8 years prior to the patient's birth. On examination, the patient had unclear speech and slow speech rate, with diminished reading comprehension and calculation abilities. Characteristic facial features of KS were noted. Breast development was observed (Tanner stage II on the right and III on the left), with pain upon deep palpation of the left nipple. Molecular genetic testing identified a heterozygous missense mutation, c.15535C>T (p.Arg5179Cys), in the KMT2D gene, confirming the diagnosis of type I Kabuki syndrome. DISCUSSION: KS is characterized by distinctive facial features: arched eyebrows, eversion of the eyelids, long palpebral fissures, a short nasal septum, a flat nasal tip, auricular deformities, a small mandible, a high palatal arch, or cleft palate. The patient exhibited a heterozygous missense mutation in the coding region of the KMT2D gene, identified as a de novo mutation. Currently, KS management primarily involves symptomatic and rehabilitative therapies.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11926138/

Record 172

pmcid: PMC12336495

pmid: 40792282

doi: 10.3389/fonc.2025.1573915

title: Case Report: Breast cancer in pregnancy with placental metastasis.

authors

- Lin Y
- Xie X
- Wang J
- Chen H
- Wan N
- Lin J

journal: Frontiers in oncology

journalAbbr: Front Oncol

year: 2025

pubdate: 2025

volume: 15

issue:

pages: 1573915

abstract: With the increasing incidence of breast malignant tumours in the population, some rare cases of breast cancer have been reported. We would like to share with you a case of placental metastasis of a malignant tumour of the breast: the patient was a young woman who was diagnosed with a breast cancer at 31 weeks of gestation. After regular follow-up, a healthy baby girl was delivered by caesarean section at 37 weeks of pregnancy. In a subsequent placenta specimen, the pathologist found distant metastasis of breast cancer to the placenta. This is rare in distant metastases of breast cancer. The patient is currently undergoing further anti-tumour therapy. Through reviewing the relevant literature, we have compiled and discussed the mechanisms related to the possible use of placenta in preventing cancer invasion and how breast cancer in pregnancy can be followed up. We hope that sharing this case will help further clinical and experimental research.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12336495/

Record 173

pmcid: PMC12728831

pmid: 41450767

doi: 10.3389/fsurg.2025.1702539

title: Case Report: Laparoscopic vaginoplasty in a case of partial androgen insensitivity syndrome and a literature review of 16 cases in China.

authors

- Qin H
- Liu H
- Liu Q
- Li B

journal: Frontiers in surgery

journalAbbr: Front Surg

year: 2025

pubdate: 2025

volume: 12

issue:

pages: 1702539

abstract: BACKGROUND: We examined the clinical features, diagnosis, treatment, and future gender choice of patients with partial androgen insensitivity syndrome (PAIS). CASE PRESENTATION: The clinical features, specialty examinations, three-stage surgical examination, and treatment process of a patient with PAIS were comprehensively reviewed. Additionally, 16 PAIS case reports were collected from Chinese databases and analyzed. Seventeen PAIS cases were included in this study. All patients presented with primary amenorrhea (17/17) and displayed specific clinical features such as a male social identity or appearance (3/17), an underdeveloped phallus or clitoromegaly (16/17), abnormal urethral development (7/17), and breast development (4/17). Twelve cases were managed as women, both surgically and with hormone replacement therapy. The patient in this case report underwent three surgical procedures involving laparoscopic examination and clitoroplasty, left orchidopexy, and laparoscopic vaginoplasty using peritoneum and an acellular dermal matrix. CONCLUSIONS: PAIS is a rare disorder of sex development (DSD) with a 46,XY karyotype and is a congenital X-linked recessive disease. Therefore, a deeper understanding of the pathogenesis of androgen

insensitivity syndrome allows more accurate diagnosis, personalized treatment, and organized follow-up of the condition, thereby avoiding gender dysphoria.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12728831/

Record 174

pmcid: PMC11981431

pmid: 40212185

doi: 10.1097/MS9.0000000000003153

title: Clinical and surgical outcomes of virginal breast hypertrophy (VBH) in adolescent females: a systematic review of case reports.

authors

- Siddiqui E
- Khalid M
- Khan MS
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journal: Annals of medicine and surgery (2012)

journalAbbr: Ann Med Surg (Lond)

year: 2025

pubdate: 2025 Apr

volume: 87

issue: 4

pages: 2286-2294

abstract: BACKGROUND: The rare medical condition known as Virginal Breast Hypertrophy causes massive adolescent breast growth that leads to the important health problems with social implications. This systematic review examines treatment procedures and psychological effects along with identifying remaining knowledge gaps regarding this disorder. METHODS: The research team executed a PRISMA-based literature search covering cases from the year 1934 to 2023, and analyzed 69 cases of VBH. The analysis included information about symptoms at diagnosis and treatment methods along with the recurrence rates and details about psychological elements. The study included patients who experienced juvenile breast hypertrophy during adolescence. RESULTS: VBH typically presented with rapid bilateral breast enlargement (91.3%) and psychological distress (71%). The mean age of onset was 11.27 years, with disease progression averaging 10 months. Menarche was documented in 43.75%. Subcutaneous mastectomy showed the lowest recurrence rate (5.8%), while reduction mammoplasty had a recurrence rate of 44.7%. Medical treatments (e.g., tamoxifen, bromocriptine) showed variable outcomes, with most patients exhibiting no hormonal abnormalities. CONCLUSIONS: Among breast cancer treatments subcutaneous mastectomy stands as the most successful method that results in few recurrences. Documentation of psychological distress created by VBH demands collaborative care strategies and extensive investigations into genetic molecular influences.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11981431/

Record 175

pmcid: PMC11813032

pmid: 39928780

doi: 10.1097/MD.0000000000041483

title: Clinical characteristics and risk factors of female precocious puberty.

authors

- Qi H
- Pu S
- Zhai H

journal: Medicine

journalAbbr: Medicine (Baltimore)

year: 2025

pubdate: 2025 Feb 7

volume: 104

issue: 6

pages: e41483

abstract: This study explores the clinical characteristics and risk factors of female precocious puberty. This retrospective study was conducted on 119 girls with precocious puberty admitted to Pu'er People's Hospital from January 2022 to November 2023 (PP group) and 119 girls with normal sexual maturity during the same period (normal group). Clinical characteristics (including laboratory indicators) and general conditions of participants in both groups were collected and compared. The risk factors of female precocious puberty were analyzed using logistic analysis. There were statistically significant differences in axillary hair growth, early menarche, vaginal mucus appearance, pubic hair growth, breast development, areola diameter, ovarian volume, uterine volume, bone age, body mass index, height, prolactin, progesterone, estradiol, follicle stimulating hormone, and luteinizing hormone levels between the 2 groups (P < .05). There were statistically significant differences in frequent use of cosmetics, polluted living environment (chemical and pesticide detergents), sleeping with the light on at night, early exposure to sexual information, consumption of fried foods > 2 times/week, consumption of snacks > 2 times/week, parental discord, consumption of off-season fruits and vegetables, consumption of domestic poultry > 7 times/week, mother's menarche < 12 years, and mother's breast development age ≤ 10 years between the 2 groups (P < .05). Logistic analysis confirmed that frequent use of cosmetics, polluted living environment (chemical and pesticide detergents), sleeping with the light on at night, early exposure to sexual information, consumption of fried foods > 2 times/week, consumption of snacks > 2 times/week, parental discord, consumption of off-season fruits and vegetables, consumption of domestic poultry > 7 times/week, mother's menarche < 12 years old, and mother's breast development age ≤ 10 years old were risk factors of female precocious puberty (P < .05). In clinical practice, targeted prevention and control can be implemented based on relevant risk factors to improve the physical and mental health of children.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11813032/

Record 176

pmcid: PMC12365484

pmid: 40843322

doi: 10.14744/nci.2024.65049

title: Clinical, hormonal and radiological features, and treatment outcomes of prolactinomas in a pediatric population.

authors

- Emeksiz HC

journal: Northern clinics of Istanbul

journalAbbr: North Clin Istanb

year: 2025

pubdate: 2025

volume: 12

issue: 3

pages: 269-276

abstract: OBJECTIVE: Prolactinoma is the most common hormone-secreting pituitary tumor in the pediatric population. Although it is less common in children and adolescents than in adults, it accounts for 50% of childhood pituitary adenomas. Data on prolactinomas in the pediatric population are still limited. In this study, the symptoms, laboratory data, radiological findings, and therapeutic outcomes of prolactinomas in children and adolescents were assessed. METHODS: This retrospective study included pediatric patients diagnosed with prolactinomas before 18 years of age, who presented at Istanbul Medeniyet University, Professor Doctor Suleyman Yalcin City Hospital during an 8-year period (August 2015 to November 2023). RESULTS: Seventeen patients (13 female; 76.4%) with prolactinoma were included. Median age at diagnosis was 14.7 years (12.2-16.1 years) in girls, 11.8 years (6.8-16.2) years) in boys. All boys and most girls (62%) had macroadenomas (≥1 cm). The most common presenting symptom was amenorrhea/oligomenorrhea in girls and, mass effects and gynecomastia in boys. The median prolactin (PRL) level was significantly higher in the macroprolactinoma group than in the microprolactinoma group (262.5 vs. 178 ng/mL; p=0.035). Cabergoline was introduced to all patients as first-line treatment and normal PRL level was achieved in 88.3% of them after a two-year treatment. One male and one female patient were unresponsive to 2 mg/week cabergoline treatment and therefore underwent transsphenoidal surgery. PRL elevation recurred in six of seven patients (86%) after the withdrawal of cabergoline treatment. CONCLUSION: A macroprolactinoma is more common in children and adolescents than a microprolactinoma in adults. Increased PRL levels, male gender and the presence of mass effects at the time of diagnosis are associated with macroprolactinomas diagnosed during childhood and adolescence. Cabergoline was highly effective in the treatment of pediatric prolactinomas. However, due to the high recurrence rate of hyperprolactinemia after withdrawal of a 2-year treatment, use of cabergoline for a longer duration (≥3 years) before the first withdrawal attempt might be beneficial to reduce the risk of recurrence in selected pediatric cases with macroprolactinoma.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12365484/

Record 177

pmcid: PMC12221001

pmid: 40601568

doi: 10.1371/journal.pone.0325380

title: Comparative analysis of skin microbiome of patients with filarial lymphedema and healthy individuals.

authors

- Manavalan S
- Pradeep D
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journal: PloS one

journalAbbr: PLoS One

year: 2025

pubdate: 2025

volume: 20

issue: 7

pages: e0325380

abstract: BACKGROUND: Lymphatic filariasis, a vector borne parasitic disease is a public health problem in the tropical region. Recurrent skin and soft tissue infections termed adenolymphangitis (ADL) is a major complication of filarial lymphedema. Understanding the changes in skin microbiome associated with this disease may provide novel insights on ADL attacks and lymphedema progression. This study investigates the changes in skin microbial flora in patients affected with filarial lymphedema. METHODS: We employed 16S rRNA gene amplicon-based metagenomic technique to profile the skin microbiome of patients with filarial lymphedema in comparison with healthy volunteers. RESULTS: There were notable differences in the bacterial flora between patients and healthy controls. Actinobacteria were under-represented in the patient group. Staphylococcus dominated both the groups, 63% in patients and 44% in controls. Samples from a few patients showed the presence of certain rare bacteria like Eremococcus and Facklamia. CONCLUSION: This pilot study applying advanced molecular tools provides insight on the changes in skin microflora associated with filarial lymphedema for the first time. Further studies are necessary for a better understanding of the role of the altered skin microbiome in frequent episodes of adenolymphangitis in patients with filarial lymphedema.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12221001/

Record 178

pmcid: PMC12041335

pmid: 40301606

doi: 10.1038/s41598-025-99834-8

title: Comparison of nipple sparing and skin sparing mastectomy with immediate reconstruction based on patient reported outcomes.

authors

- Song Y
- Wang L
- Huang W
- Guo S
- Wu X
- Zheng H

journal: Scientific reports

journalAbbr: Sci Rep

year: 2025

pubdate: 2025 Apr 29

volume: 15

issue: 1

pages: 14989

abstract: Nipple-sparing mastectomy (NSM) and skin-sparing mastectomy (SSM) are widely accepted surgical options for breast cancer patients undergoing immediate reconstruction. However, the impact of preserving the nipple-areolar complex on patient-reported outcomes (PROs) remains uncertain, particularly in clinically matched settings. This retrospective study included patients who underwent unilateral NSM or SSM followed by immediate implant-based breast reconstruction at Hubei Cancer Hospital between January 2022 and January 2024. Patients were propensity score matched (2:1) based on age, body mass index, preoperative breast size, neoadjuvant chemotherapy, implant plane, and postmastectomy radiation therapy. PROs were assessed using the BREAST-Q (version 2.0) and the decision regret scale. Univariate and multivariate linear regression analyses were conducted to identify predictors of satisfaction and regret. A total of 87 patients were included after matching (NSM: n = 58; SSM: n = 29). NSM patients reported significantly higher satisfaction with breasts (mean 58.1 vs. 52.9, P = 0.038) and sexual well-being (mean 52.4 vs. 38.9, P = 0.035). Although psychosocial well-being and decision regret showed favorable trends in the NSM group, differences were not statistically significant. Multivariate analysis revealed that reduction in breast size was significantly associated with

decreased satisfaction and psychosocial well-being, while increased breast size was linked to greater decision regret. No significant differences were observed in complication rates between groups. Notably, none of the SSM patients received nipple reconstruction during follow-up. NSM was associated with greater satisfaction in breast and sexual well-being compared to SSM in matched breast cancer patients undergoing implant-based reconstruction. These findings support prioritizing NSM when oncologically feasible and highlight the potential role of delayed nipple reconstruction and expectation management in improving postoperative satisfaction for SSM patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12041335/

Record 179

pmcid: PMC12200214

pmid: 40575612

doi: 10.1097/GOX.0000000000006913

title: Complications and Satisfaction After Adolescent Breast Reduction for Juvenile Macromastia: Systematic Review and Meta-analysis.

authors

- Halawani IR
- Alalawi S
- Alyamani S
- Alhithlool AW
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journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2025

pubdate: 2025 Jun

volume: 13

issue: 6

pages: e6913

abstract: BACKGROUND: Juvenile macromastia is a rare condition of significant breast enlargement in adolescents. Reduction mammoplasties offer relief, but data on complications in adolescents are rare as opposed to data on adults. We reviewed the outcomes, complications, recurrence, and patient satisfaction after reduction mammoplasties in adolescents. METHODS: Following the Preferred Reporting Items for Systematic Reviews and Meta-Analyses guidelines, we conducted a thorough search across various electronic databases for "juvenile macromastia" and "breast reduction." Only studies on women diagnosed with juvenile macromastia before 21 years of age who underwent breast reduction surgery were included. The methodological index for nonrandomized studies was used to assess study quality. RESULTS: This meta-analysis pooled data from 11 studies. The overall pooled postoperative complication rate, based on random-effects models, was 17.5% (95% confidence interval: 9.7%-29.5%). The recurrence rate was 15.6% (95% confidence interval: 8.5%-26.9%), ranging from 0% to 52.9%. Complication rates varied widely across studies, with wound dehiscence, hematoma, and infection being the most common. Severe complications, such as nipple necrosis, were rare. Low publication bias was observed for postoperative complications, but potential bias was noted for recurrence outcomes. CONCLUSIONS: The findings emphasize the need for standardized reporting and long-term follow-up to improve the reliability of pooled estimates and to guide clinical decision-making. The high recurrence risk emphasized the need for individualized surgical approaches and careful management of risk factors, such as

obesity and smoking, to improve outcomes. Despite the variability, the benefits of surgery generally outweighed the risks, with high patient satisfaction reported in the included studies.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12200214/

Record 180

pmcid: PMC12210177

pmid: 40606934

doi: 10.4254/wjh.v17.i6.108096

title: Correlation between gynecomastia and endocrine regulation in patients with metabolic dysfunction-associated fatty liver disease: A cross-sectional study.

authors

- Zhang MH
- Meng N
- Zhang KH
- Yu JK
- Huang CH
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journal: World journal of hepatology

journalAbbr: World J Hepatol

year: 2025

pubdate: 2025 Jun 27

volume: 17

issue: 6

pages: 108096

abstract: BACKGROUND: Metabolic dysfunction-associated fatty liver disease (MASLD) is a chronic liver disease characterized by hepatocellular steatosis, which is closely related to metabolic syndrome, with annually increasing morbidity in recent years. Gynecomastia (GYN), an abnormal proliferation of breast tissue in males, is common in males with sex hormone imbalance. Currently, there is insufficient research on the morbidity of GYN and its correlation among MASLD patients. AIM: To investigate the prevalence of GYN and its associated clinical features in patients with MASLD and determine the prevalence of GYN in patients with MASLD and analyze the predictive effect of sex hormones on GYN using receiver operating characteristic (ROC) curves. METHODS: A cross-sectional study was conducted in a tertiary care hospital. Among them, 997 patients met the inclusion criteria and underwent breast ultrasonography to determine the presence of GYN. Anthropometric data, laboratory test data [estradiol (E2), androgens, liver function, glucose, lipids, low-density lipoprotein, high-density lipoprotein, creatinine, and uric acid, etc.], as well as information on medical history, severity of fatty liver (mild, moderate, and severe), and duration of the disease were collected. Package for the Social Sciences version 27 and R software (version 4.3.1) were used for data analysis. RESULTS: The prevalence of GYN increased with the severity of fatty liver (27.6% for mild, 33.5% for moderate, and 58% for severe, $P < 0.001$); compared with non-GYN patients, GYN patients were older (54.11 ± 9.71 years vs 47.89 ± 9.92 years, $P < 0.001$), with significantly higher E2 levels, higher estrogen to androgen ratio ($P < 0.001$) and significantly lower androgen levels ($P < 0.001$). In ROC curve analysis, the combined model of testosterone and E2 had a high diagnostic value in predicting GYN in MASLD patients, surpassing a single indicator. CONCLUSION: The presence of GYN may suggest more severe metabolic abnormalities in patients with MASLD. Therefore, early recognition of GYN may be crucial for early intervention in metabolic syndrome and endocrine abnormalities in patients with MASLD.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12210177/

Record 181

pmcid: PMC12622666

pmid: 41239602

doi: 10.1097/MD.0000000000045872

title: Cross-tissue transcriptome-wide association and Mendelian randomization identify RALB as a susceptibility gene for breast hypertrophy.

authors

- Gu J
- Chen J
- Hu Z
- Luo H

journal: Medicine

journalAbbr: Medicine (Baltimore)

year: 2025

pubdate: 2025 Nov 14

volume: 104

issue: 46

pages: e45872

abstract: Breast hypertrophy is a pathological condition characterized by abnormal enlargement of the breasts due to excessive glandular and adipose tissue proliferation. It is associated with physical discomfort and reduced quality of life. However, its underlying genetic basis remains largely unexplored. We conducted a cross-tissue transcriptome-wide association study (TWAS) using the UTMOST framework, integrating expression quantitative trait locus data from GTEx and GWAS summary statistics from the FinnGen database (7272 cases, 258,508 controls). Additional validation was performed using single-tissue TWAS (FUSION), gene-level association testing (Multi-marker Analysis of GenoMic Annotation (MAGMA)), fine-mapping (FOCUS), and conditional and joint analyses. Key candidate genes were subjected to Mendelian randomization (MR) and Bayesian colocalization analyses to investigate causality. A total of 44 genes were significantly associated with breast hypertrophy in cross-tissue TWAS (false discovery rate < 0.05), and 4 were replicated in whole blood via FUSION. Fine-mapping identified RAS Like Proto-Oncogene B (RALB) as the most probable causal gene (PIP = 0.98), supported by MAGMA, conditional analyses, and colocalization (PPH4 = 0.922). Mendelian randomization confirmed a significant causal relationship between elevated RALB expression and increased risk of breast hypertrophy (odds ratio = 1.26; 95% confidence interval: 1.17-1.35; P < .001). Our integrative genomic analyses identified RALB as a robust susceptibility gene for breast hypertrophy. These findings provide novel insights into the genetic etiology of breast hypertrophy and may facilitate future development of molecular diagnostics or targeted therapies.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12622666/

Record 182

pmcid: PMC12558728

pmid: 41163921

doi: 10.3389/fped.2025.1663626

title: Cross-union surgery for tibiofibular fractures in McCune-Albright syndrome: a case report and literature review.

authors

- Zhu H

- Wang Y
- Zhuang H
- Zheng P
- Tang Y

journal: Frontiers in pediatrics

journalAbbr: Front Pediatr

year: 2025

pubdate: 2025

volume: 13

issue:

pages: 1663626

abstract: INTRODUCTION: McCune-Albright syndrome (MAS) is a rare disease. MAS manifests in the skeletal system as Fibrous dysplasia (FD). In MAS patients, bone lesions often result in fractures under weight-bearing forces during childhood, and pseudarthrosis formation is highly likely after fractures. There is currently no ideal treatment for fractures in children with MAS. This case study presents the innovative application of the cross-union technique in treating a 14-year-old female MAS patient with a distal tibial and fibular fracture nonunion. METHODS: The patient had a history of conservative treatment with persistent symptoms, including an inability to bear weight and walk independently. A "cross-union" procedure was performed using a Fassier-Duval rod, locking plate, and K-wire fixation. RESULTS: At the final follow-up 12 months postoperatively, x-rays revealed a stable "cross-union" bone connection at the distal ends of the tibia and fibula, with no occurrence of refracture or other complications. The Radiographic Union Score for Tibial Fractures (RUST) was 12, and the Olerud Molander Ankle Score was 60. CONCLUSION: We successfully applied the "cross-union" technique to a MAS child with nonunion after a distal tibiofibular fracture. We look forward to further improvement and promotion of the "cross-union" technology in the future, bringing new hope for treating fractures in MAS/FD children.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12558728/

Record 183

pmcid: PMC12425455

pmid: 40925326

doi: 10.1016/j.devcel.2025.06.032

title: Defining breast epithelial cell types in the single-cell era.

authors

- Gray GK
- Carlson EG
- Lev T
- Marshall B
- Reed AD
- Sánchez-Covarrubias AP
- Twigger AJ
- Puig-Barbe A
- Thennavan A
- Omotoso A
- Murrow LM
- Chatterjee D
- He S

- Pensa S
- Aevermann B
- Tavares NK
- Chen N
- Hilton JA
- Blake K
- Liu Y
- Phong K
- Gartner ZJ
- Lawson DA
- Swarbrick A
- Dos Santos CO
- George SHL
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- LaBarge MA
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- Navin N
- Kessenbrock K
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journal: Developmental cell

journalAbbr: Dev Cell

year: 2025

pubdate: 2025 Sep 8

volume: 60

issue: 17

pages: 2218-2236

abstract: Single-cell studies on breast tissue have contributed to a change in our understanding of breast epithelial diversity that has, in turn, precipitated a lack of consensus on breast cell types. The confusion surrounding this issue highlights a possible challenge for advancing breast atlas efforts. In this perspective, we present our consensus on the identities, properties, and naming conventions for breast epithelial cell types and propose goals for future atlas endeavors. Our proposals and their underlying thought processes aim to catalyze the adoption of a shared model for this tissue and to serve as guidance for other investigators facing similar challenges.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12425455/

Record 184

pmcid: PMC12342733

pmid: 40163681

doi: 10.1093/jnci/djaf088

title: Development of a multi-institutional dataset to validate a novel inflammatory breast cancer diagnostic score.

authors

- Lynce F

- Niman SM
- Kai M
- Ryan S
- Troll E
- Li L
- Miller KD
- Jagsi R
- Mason G
- Overmoyer BA
- Le-Petross HTC
- Nakhlis F
- Krishnamurthy S
- Harrison BT
- Sun SX
- Yeh ED
- Bellon JR
- Warren LE
- Stauder MC
- Regan MM
- Woodward WA

journal: Journal of the National Cancer Institute

journalAbbr: J Natl Cancer Inst

year: 2025

pubdate: 2025 Aug 1

volume: 117

issue: 8

pages: 1583-1592

abstract: BACKGROUND: Susan G. Komen, the Inflammatory Breast Cancer (IBC) Research Foundation, and the Milburn Foundation convened patient advocates, clinicians, and researchers to propose novel quantitative scoring rubrics for IBC diagnosis. In this study, we developed a multi-institutional clinical dataset to test and validate the proposed scoring system. METHODS: IBC (N = 988) and non-IBC (N = 332) cases were identified at 2 institutions with dedicated multidisciplinary IBC programs. The non-IBC cohort included consecutive cT4b and cT4c patients. Standard operating procedures (SOPs) were developed for all ambiguous findings and languages. Three different methods were used for the imputation of missing data, resulting in 3 separate datasets. The sensitivity, specificity, and area under the receiver operator characteristic curve (AUC-ROC) were used to assess the discrimination of the proposed scoring rubric. RESULTS: The distribution of "true IBC" cases was 19.7% very likely IBC, 49.1% strong possibility of IBC, 0.4% weak possibility of IBC, 0.1% very unlikely IBC, and 30.7% unknown; corresponding groupings for true non-IBC cases were 0.6% very likely IBC, 51.8% strong possibility of IBC, 9.9% weak possibility of IBC, 2.1% very unlikely IBC, and 35.5% unknown. The AUC-ROC values for missing data imputation methods were similar (0.83-0.84); exploratory score refinement improved the AUC-ROC to 0.88-0.89. CONCLUSION: Using the largest multi-institutional IBC clinical database to date, the score has been validated and is available for clinical use at https://www.komen.org/ibc-calc to assist health-care providers and their patients in IBC diagnosis. Exploratory score refinement demonstrates the potential to increase specificity; however, any change requires separate validation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12342733/

Record 185

pmcid: PMC12063851

pmid: 40343978

doi: 10.1371/journal.pone.0323549

title: Development of a simplified prediction model for diagnosing progressive central precocious puberty using clinical and pelvic ultrasound parameters.

authors

- Song K
- Lee E
- Lee HS
- Lee H
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- Shin HJ

journal: PloS one

journalAbbr: PLoS One

year: 2025

pubdate: 2025

volume: 20

issue: 5

pages: e0323549

abstract: This study aimed to explore the predictive value of clinical and pelvic ultrasound parameters for diagnosing central precocious puberty (CPP) and to establish a clinically useful simplified prediction model to differentiate progressive CPP (P-CP) from nonprogressive precocious puberty (N-PP). Girls aged <9 years with secondary sexual development who underwent a gonadotropin-releasing hormone stimulation test and pelvic ultrasound between September 2020 and November 2023 were retrospectively included and divided into the P-CP and N-PP groups. Logistic regression analysis was used to determine the significant parameters and develop prediction models. The diagnostic performance of the models was compared using the area under the receiver operating characteristic curve (AUC) analysis and the Delong method. The continuous net reclassification improvement (cNRI) and absolute integrated discrimination improvement (IDI) were used to determine the additive effects of ultrasound parameters. A nomogram scoring system was constructed based on a simplified model to predict the probability of developing P-CP. A total of 109 girls were included, with 64 (58.7%) in the P-CP group. Age, bone age, height, height minus midparental height, basal luteinizing hormone (LH), follicle-stimulating hormone, estradiol, insulin-like growth factor-I, Tanner stage, and cervical and fundus width were significant parameters for the diagnosis of P-CP. The models with ultrasound parameters yielded significantly higher cNRI and IDI values than the models without ultrasound parameters. The simplified model was composed of basal LH, estradiol, and fundus width that showed an AUC value of 0.93 (95% confidence interval: 0.88-0.98) with a cutoff value of 16. In conclusion, adding pelvic ultrasound parameters to traditional clinical results has an additive effect on P-CP screening. A simplified predictive model is effective for CPP screening in real-world clinics. These findings highlight the potential of the prediction model to overcome the limitations of the classical diagnostic approach for CPP in children.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12063851/

Record 186

pmcid: PMC12703463

pmid: 41384863

doi: 10.1530/EC-25-0642

title: Diagnostic accuracy of the triptorelin stimulation test for central precocious puberty in girls.

authors

- Chioma L
- Bizzarri C
- Longobardi M
- Mainieri F
- Porzio O
- D'Agostini M
- Paone L
- Bottaro G
- Cianfarani S

journal: Endocrine connections

journalAbbr: Endocr Connect

year: 2025

pubdate: 2025 Dec 1

volume: 14

issue: 12

pages:

abstract: Central precocious puberty (CPP) results from premature activation of the hypothalamic-pituitary-gonadal axis. The GnRH stimulation test remains the diagnostic gold standard, especially in girls with undetectable basal LH levels. However, native GnRH is expensive and often unavailable. Rapid-acting subcutaneous triptorelin has been proposed as a reliable alternative. This retrospective study evaluated the diagnostic accuracy of subcutaneous triptorelin compared with intravenous gonadorelin and investigated the optimal timing for LH peak assessment. A total of 341 girls referred for suspected precocious puberty were evaluated: 102 underwent the triptorelin test and 239 the gonadorelin test, with gonadotropins measured by electrochemiluminescence assay at baseline and after stimulation. Based on clinical, radiological, and laboratory criteria, 143 girls were diagnosed with CPP and 198 with non-progressive thelarche (NPT). Triptorelin elicited significantly higher FSH peaks than gonadorelin, while LH peaks were comparable; consequently, FSH/LH ratios were higher after triptorelin. ROC analysis identified an optimal diagnostic LH peak cut-off of 7.14 IU/L following triptorelin administration (sensitivity 94%, specificity 96%; AUC 0.985), while a threshold of 4.7 IU/L was observed after gonadorelin (sensitivity 100%, specificity 87%; AUC 0.982). Peak LH occurred predominantly at 180 min after triptorelin in both CPP and NPT groups (78 and 90%, respectively). Despite the limitations of its retrospective and non-parallel design, this large cohort study demonstrates that subcutaneous triptorelin provides excellent diagnostic accuracy, comparable to gonadorelin. These findings support triptorelin as a reliable and accessible alternative for CPP diagnosis and contribute to the standardization of diagnostic protocols in clinical practice. PLAIN LANGUAGE SUMMARY: This study tested triptorelin stimulation as an alternative to the standard gonadorelin test to diagnose precocious puberty in girls. Triptorelin proved equally accurate and reliable, especially when hormone levels are measured 3 h after injection, offering a practical diagnostic tool in clinical practice.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12703463/

Record 187

pmcid: PMC12304955

pmid: 40881099

doi: 10.5409/wjcp.v14.i3.102741

title: Diagnostic and management challenges in a partially infarcted borderline phyllodes tumor in an adolescent female: A case report and review of literature.

authors

- Suschana E
- Sta Ines FM
- Manrai P
- Koelliker S
- Gass JS
- Tseng YA

journal: World journal of clinical pediatrics

journalAbbr: World J Clin Pediatr

year: 2025

pubdate: 2025 Sep 9

volume: 14

issue: 3

pages: 102741

abstract: BACKGROUND: Fibroadenomas (FA) and phyllodes tumors (PT) are fibroepithelial neoplasms and are difficult to differentiate radiographically and histologically. We present a partially infarcted borderline PT in an adolescent with rapid tumor enlargement within 24 hours. Tumor infarction made the diagnostic work-up difficult. Complete surgical excision is the standard of care for PTs. There is controversy regarding margin re-excision for borderline PTs. In this report, we discuss the diagnostic challenges of PT and the evolving concept of margin status on PT recurrence rate. CASE SUMMARY: A 14-year-old healthy female with no medical history presented with a painful right breast mass with no nipple discharge, trauma, or skin findings. The mass showed rapid enlargement over 24 hours, prompting a workup with ultrasound and core needle biopsy. The initial biopsy was limited due to large areas of infarction. Based on the scant viable tissue and considering the patient's age, the mass was favored to be a juvenile FA. The patient underwent excision of the mass. Final pathology confirmed a borderline PT with positive surgical margins. The patient underwent margin re-excision, which did not show any residual tumor. At the 6-month post-op visit, there was a mass-forming lesion on the breast ultrasound. Subsequent core needle biopsy showed benign breast parenchyma with scar formation. The primary goal of evaluation in pediatric breast masses is to do no harm. However, rapidly growing and symptomatic masses require a more extensive work-up including biopsy and surgical excision. We present a rapidly growing breast mass in a 14-year-old female which was diagnosed as a borderline PT on her excision specimen. The mass rapidly enlarged over 24 hours. The initial biopsy pathology was limited due to a large area of infarction. The patient underwent excision of the mass. Final pathology confirmed a borderline PT that extended into the surgical margin, resulting in an additional re-excision procedure. Accurate diagnosis prior to surgical intervention is crucial to avoid additional procedures. Although histological morphology remains the gold standard for diagnosis, immunohistochemistry and molecular studies have recently shown to improve the accuracy of diagnosis of PTs. Long-term clinical and pathologic follow-up of PTs in adolescent patients should be collectively studied to examine whether our current diagnostic criteria for PT can reliably predict tumor behavior in this age group. CONCLUSION: Accurate diagnosis of PTs requires surgical excision. Tumor infarction may lead to rapid tumor enlargement, hindering the correct diagnosis. More research is needed on margin status and recurrence rate, especially in adolescent patients, to help establish the best possible care for this age group.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12304955/

Record 188

pmcid: PMC12469236

pmid: 41008766

doi: 10.3390/diagnostics15182395

title: Diagnostic and Therapeutic Challenges in Pseudoangiomatous Stromal Hyperplasia (PASH) of the Breast in a 14-Year-Old Girl: A Case-Based Review.

authors

- Sosnowska-Sienkiewicz P
- Mańkowski P
- Januszkiewicz-Lewandowska D

journal: Diagnostics (Basel, Switzerland)

journalAbbr: Diagnostics (Basel)

year: 2025

pubdate: 2025 Sep 20

volume: 15

issue: 18

pages:

abstract: Introduction: Pseudoangiomatous stromal hyperplasia (PASH) is a benign breast lesion characterized by stromal myofibroblast proliferation forming slit-like pseudoangiomatous spaces. Although most frequently diagnosed in premenopausal women, it has also been reported in adolescent girls, where it may present as a rapidly enlarging mass that mimics fibroadenoma or phyllodes tumor. The pathogenesis is thought to be hormonally influenced, particularly by progesterone, with a possible role for estrogen. Case Report: We report the case of a 14-year-old girl who presented with a painless, rapidly growing mass in the left breast, first noticed approximately six months earlier. Clinical examination revealed a mobile lesion about 10 cm in diameter without skin changes, lymphadenopathy, or nipple discharge. Ultrasound and MRI demonstrated a large, well-circumscribed solid tumor (10.4 × 11.2 × 4.2 cm 3) displacing normal breast tissue but without infiltration; both were classified as BI-RADS 4. Given the tumor size, diagnostic uncertainty, and potential risk of a non-representative core needle biopsy, a decision was made to proceed with primary radical excision. The mass was completely removed with preservation of the glandular tissue. Histopathology confirmed PASH, described macroscopically as a solid, gray-yellow, encapsulated tumor and microscopically as slit-like spaces lined by spindle cells (CD34+, CD31-). Postoperatively, the breast gradually regained symmetry with the contralateral side, and at 14 months of follow-up, no recurrence was observed. Conclusions: PASH, although benign, may present as a large breast tumor in adolescents and clinically mimic both benign and malignant lesions. Histological evaluation based on an adequately performed biopsy is crucial for accurate diagnosis. Complete excision with capsule preservation is recommended to minimize the risk of recurrence. In adolescents, a watchful waiting approach after surgery may be beneficial, as breast tissue often remodels and regains symmetry spontaneously, reducing the need for reconstructive procedures. This case underscores the importance of individualized diagnostic and therapeutic strategies in managing rare benign breast lesions in pediatric patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12469236/

Record 189

pmcid: PMC12399815

pmid: 40900764

doi: 10.1177/2050313X251370506

title: Discectomy with spinal anesthesia in the third trimester of pregnancy: A case report and literature review.

authors

- Mehrafshan A
- Hajilo P
- Samiee F

journal: SAGE open medical case reports

journalAbbr: SAGE Open Med Case Rep

year: 2025

pubdate: 2025

volume: 13

issue:

pages: 2050313X251370506

abstract: Low back pain is common during pregnancy, but symptomatic lumbar disc herniation in this period is particularly rare. We report a case of lumbar disc herniation in a 35-year-old woman at 32 weeks of pregnancy, who experienced persistent symptoms despite 6 weeks of rest and medication. Muscle strength in the right L5 nerve root declined to grade 4, and intravenous analgesia was ineffective. After thorough discussions with the patient and her family, a discectomy was performed under spinal anesthesia. Following surgery, the patient's pain decreased, the Lasegue test became negative, and muscular strength returned to normal 2 weeks post-operation. At 39 weeks of gestation, a cesarean section was performed due to functional dystocia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12399815/

Record 190

pmcid: PMC12731835

pmid: 41451407

doi: 10.1016/j.fsisyn.2025.100652

title: Documentation of biological sex and gender in missing person investigations.

authors

- Winter KA
- Birkmann-Little C

journal: Forensic science international. Synergy

journalAbbr: Forensic Sci Int Synerg

year: 2025

pubdate: 2025 Dec

volume: 11

issue:

pages: 100652

abstract: This paper will discuss the current understanding of biological sex and gender, its limited recording and the subsequent implications on missing persons investigations. The current understanding of biological sex and gender on a binary is inaccurate and can be seen to be insufficient when recording biological sex and gender information about missing and unknown individuals. This understanding ignores intersex, trans and gender diverse individuals and consigns them to either fit the 'normal' binary definitions or else places them along a binary-based spectrum or in a 'indeterminate' category. This is unacceptable, unethical and harmful. Instead, biological sex and gender should not be seen as binary but diverse. The idea of sexual dimorphism should be replaced with an understanding of sexual polymorphism, and biological sex and gender should no longer be defined on a male-female binary, spectrum or scale, but explored on a multifactorial polygon and recorded as such. Cognitive and contextual biases have restricted gender and sexual based inclusion in the long-term recording as the binary has long been considered the 'normal'. This has become a structural way to continue the violence against LGBTQIA2+ individuals, in addition to cis individuals, because everyone is affected by the imposed binary. This paper will highlight the issues associated with this 'normal', expose the limitations of a biological sex and gender binary in the recording forms (from a global sample of 170 forms), and suggest changes to further progress the understanding of polymorphism to improve inclusive recording in missing persons investigations.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12731835/

Record 191

pmcid: PMC12653644

pmid: 41303162

doi: 10.3390/jcm14228125

title: Drawbacks of Olanzapine Therapy: An Emphasis on Its Metabolic Effects and Discontinuation.

authors

- Oruch R
- Rajab HA
- Elderbi MA
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- Fasmer OB
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journal: Journal of clinical medicine

journalAbbr: J Clin Med

year: 2025

pubdate: 2025 Nov 17

volume: 14

issue: 22

pages:

abstract: Radical drug therapy for schizophrenia is usually hard to achieve with one currently available antipsychotic agent. Indeed, it is the negative symptoms of this morbidity that are a dilemma to neutralize. Most of the first-generation agents can deal with the positive symptoms of the disease to a convincing degree, but not with its negative symptoms. The creation of so-called second-generation agents aimed to treat the negative symptoms, as these invisible barriers are the real reasons that isolate psychotic individuals and hinder their integration into society. Unfortunately, these newly designed drugs, including OLZ, turned out to induce different categories of undesired effects; the most embarrassing among them are the metabolic drawbacks, such as insulin resistance, weight gain, and other subcategories of metabolic consequences. Antagonism induced at certain receptors, particularly 5-HT2C and histamine H1 receptors, is implicated particularly in these metabolic adverse effects. The choice of antipsychotics (APCs) should be tailored separately for each case, as each patient responds variably to each neuroleptic. This possibility exists due to the abundant alternatives within the currently available APC medications. This work aims to discuss the reasons behind these undesired metabolic effects, how to deal with them, how to choose the appropriate agent for each psychotic case, and how to manage intoxication using olanzapine. To address these inquiries, we carefully selected 154 relevant studies, including robust meta-analyses, from the past 20 years and analyzed them in this work.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12653644/

Record 192

pmcid: PMC12374115

pmid: 40842264

doi: 10.1177/23247096251367692

title: Early Manifestation of McCune-Albright Syndrome in a 32-Month-Old Female: A Rare Case Report.

authors

- Makhlouf TZ
- Mahareeq MS

- Abumazen A
- Manasrah T
- Badareen M
- Zighan MS

journal: Journal of investigative medicine high impact case reports

journalAbbr: J Investig Med High Impact Case Rep

year: 2025

pubdate: 2025 Jan-Dec

volume: 13

issue:

pages: 23247096251367692

abstract: McCune-Albright syndrome (MAS) is a rare disorder, occurring in 1 in 100 000 to 1 in 1 000 000 live births, caused by post-zygotic somatic mutations in the GNAS gene. This leads to fibrous dysplasia (FD), café-au-lait (CAL) skin pigmentation, and hyperfunctioning endocrinopathies. We present a 32-month-old girl with recurrent vaginal bleeding, bilateral breast enlargement, and multiple irregular CAL spots crossing the midline. Imaging revealed ovarian cysts and skeletal lesions consistent with FD. Laboratory findings indicated gonadotropin-independent precocious puberty. A clinical diagnosis of MAS was made, and genetic testing was deemed unnecessary. MAS presents with variable severity, and early diagnosis requires clinical recognition of its hallmark features. Genetic testing can support the diagnosis, though its reliability may vary due to mosaicism. Management is symptomatic, focusing on controlling endocrine dysfunction and minimizing skeletal complications. Emerging therapies offer promise, but no definitive cure exists. Early recognition and management are crucial for optimizing outcomes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12374115/

Record 193

pmcid: PMC12488646

pmid: 41048943

doi: 10.3389/fmed.2025.1574364

title: Efavirenz-induced grade III gynecomastia in an HIV-positive male patient: a case report and literature review.

authors

- Zhou L
- Luo Y
- Liao W
- Yang D
- Hu W

journal: Frontiers in medicine

journalAbbr: Front Med (Lausanne)

year: 2025

pubdate: 2025

volume: 12

issue:

pages: 1574364

abstract: BACKGROUND: Efavirenz, a non-nucleoside reverse transcriptase inhibitor, is a preferred drug for the treatment of HIV-positive patients. However, one of its adverse effects is gynecomastia, which not only causes psychological distress to patients, affecting their daily lives and work, but also may undermine their treatment adherence. Although numerous studies have documented cases of gynecomastia in HIV-positive patients after antiretroviral therapy (ART), severe cases of gynecomastia are rarely reported. PATIENT PRESENTATION: A 29-year-old male patient initiated a triple therapy regimen of efavirenz, lamivudine, and tenofovir disoproxil fumarate after being diagnosed with HIV. Two years later, he experienced progressive enlargement of the bilateral breasts but did not report the symptoms to his physician. After 6 years of ART, the patient developed grade III gynecomastia in both breasts, accompanied by significant psychological distress, prompting a visit to the breast surgery department. After excluding other potential drug adverse effects or underlying diseases, the patient's gynecomastia was attributed to efavirenz. Owing to severe enlargement of the breasts, the patient underwent bilateral mastectomy, successfully recovered and was discharged from the hospital. Postoperative follow-up revealed significant improvement in the patient's anxiety and depressive symptoms. CONCLUSION: Gynecomastia represents a nonnegligible adverse effect of efavirenz that clinicians must recognize and manage promptly. Prior to initiating efavirenz therapy, it is imperative to thoroughly inform patients about potential adverse reactions, emphasizing the importance of seeking medical attention promptly should any side effects arise. Early detection and management of gynecomastia can mitigate its psychological impact and ensure continued adherence to ART.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12488646/

Record 194

pmcid: PMC12144786

pmid: 40483448

doi: 10.1186/s12891-025-08544-9

title: Effects of a single session of in-person and remote strength training on low back pain and disability outcomes in pregnant women: a randomized and controlled crossover clinical trial.

authors

- Pires VJ
- Miranda C
- Fraga BS
- Führ YB
- Silva AME
- Kanitz AC

journal: BMC musculoskeletal disorders

journalAbbr: BMC Musculoskelet Disord

year: 2025

pubdate: 2025 Jun 7

volume: 26

issue: 1

pages: 573

abstract: BACKGROUND: The objective of the present study is to evaluate the effects of a single session of in-person and remote strength training on the outcomes of low back pain, functional disability, and enjoyment in pregnant women. METHODS: The methodology used was an approved controlled and randomized clinical trial by the Research Ethics Committee and registered in the Brazilian Clinical Trials Registry. The study involved seventeen pregnant women undergoing one in-person strength training session (IS), one remote strength training session (RS), and one control session (CS) in a randomly determined order. The team conducted assessments using questionnaires for low back pain, functional disability, and enjoyment. We compared the Generalized Estimating Equations (GEE) method with a Bonferroni complementary test ($\alpha = 0.05$) and the

Mann-Whitney U test. RESULTS: Reduced values of low back pain were found after the session for the SUM item in the in-person (Δ-2.06), remote (Δ-1.59), and control (Δ-2.17) sessions, as well as for the INTENSITY item in the in-person (Δ-0.23), remote (Δ-0.12), and control (Δ-0.3) sessions. Reductions in disability values 24 h after the session were observed for the remote (Δ-0.76) and control (Δ-1.59) sessions. Additionally, positive enjoyment values were noted after in-person (117) and remote (119) sessions. CONCLUSION: It is concluded that a strength training session can potentially improve or maintain the acute low back pain and disability scale without causing an increase in low back pain symptoms. Furthermore, it can promote a high level of enjoyment, regardless of whether it is conducted in person or remotely. Health education strategies, such as those proposed by the control session in this study, are also beneficial.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12144786/

Record 195

pmcid: PMC12369770

pmid: 40851946

doi: 10.1097/MS9.0000000000003370

title: Effects of vitamin supplementation on related symptoms in women with endometriosis: a systematic review and meta-analysis.

authors

- Pan B
- Bi B
- Ruan F
- Jiang Y

journal: Annals of medicine and surgery (2012)

journalAbbr: Ann Med Surg (Lond)

year: 2025

pubdate: 2025 Jul

volume: 87

issue: 7

pages: 4404-4413

abstract: BACKGROUND: Endometriosis is a gynecological disease with high occurrence, and symptoms include dysmenorrhea, infertility, and dyspareunia, which seriously affect women's quality of life. Antioxidants are beneficial in improving the symptoms of endometriosis, but there are still differences in their results. The aim of this study was to comprehensively evaluate the effects of vitamin supplementation (vitamin C, vitamin E, and vitamin D) on the related symptoms in women with endometriosis. METHODS: Electronic databases including PubMed, Web of Science, Cochrane Library, EMBASE, China National Knowledge Infrastructure, and Wanfang Data were searched up to June 2024, and the randomized controlled trials were included to evaluate the effects of vitamins on endometriosis symptoms. The primary outcomes included the scores of dysmenorrhea, pelvic pain, and dyspareunia. RESULTS: Nine studies involving 562 patients were included in this meta-analysis. Compared with the placebo group, vitamin E supplementation significantly alleviated the pelvic pain in patients with endometriosis [$P < 0.00001$, standardized mean difference (SMD) = -1.63, confidence interval (CI) = -1.98 to -1.29], but no significant differences were observed in vitamin D monotherapy or vitamin C in combination with vitamin E groups ($P = 0.13$, SMD = 0.32, CI = -0.10 to 0.75; $P = 0.10$, SMD = -0.98, CI = -2.15 to 0.18). However, vitamin E and vitamin D monotherapy or vitamin C in combination with vitamin E did not obviously improve dysmenorrhea ($P = 0.14$, SMD = -0.23, CI = -0.52 to 0.07; $P = 0.56$, SMD = -0.12, CI = -0.51 to 0.28; $P = 0.35$, SMD = -0.34, CI = -1.06 to 0.37) and dyspareunia ($P = 0.72$, SMD = 0.05, CI = -0.24 to 0.35; $P = 0.10$, SMD = -0.37, CI = -0.82 to 0.07; $P = 0.70$, SMD = 0.06, CI = -0.24 to 0.36; total $P = 0.81$, SMD = -0.02, CI = -0.22 to 0.17). In addition, vitamin D monotherapy or vitamin C in combination with vitamin E could significantly reduce the level of malondialdehyde in the serum of women with endometriosis. Moreover, no significant side

effects occurred. CONCLUSIONS: When taking vitamin E, vitamin D, or vitamin C combined with vitamin E to improve endometriosis-related symptoms, only vitamin E supplementation is beneficial to alleviate pelvic pain in women with endometriosis, and none of these vitamins can relieve endometriosis-related dysmenorrhea and dyspareunia. This study will provide a clinical reference for the treatment of endometriosis-related symptoms.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12369770/

Record 196

pmcid: PMC12359290

pmid: 40823723

doi: 10.1111/jocd.70411

title: Efficacy and Safety of Oral Spironolactone for Women With Acne Vulgaris: A Systematic Review and Meta-Analysis of Randomized Placebo-Controlled Trials With Trial Sequential Analysis.

authors

- Ghanem L
- Kirmani N
- De León Fernández N
- Palacios-Ortiz MP
- Rodríguez-Parra JD
- Rusu CA

journal: Journal of cosmetic dermatology

journalAbbr: J Cosmet Dermatol

year: 2025

pubdate: 2025 Aug

volume: 24

issue: 8

pages: e70411

abstract: BACKGROUND: Adult acne vulgaris is a chronic inflammatory skin condition that primarily affects females. Initial management includes topical and oral medications, but important limitations include ineffectiveness, nonadherence, and adverse effects. Spironolactone has shown good results in off-label acne management. AIMS: We aim to conduct a systematic review and meta-analysis exploring the safety and efficacy of oral spironolactone for females with acne. METHODS: We searched PubMed, Embase, and Cochrane for randomized controlled trials (RCTs) comparing oral spironolactone in women with acne to placebo. The primary endpoint was the objective assessment of acne improvement. Secondary endpoints included subjective assessment and adverse events. Statistical analysis was performed using Review Manager 5.4. Heterogeneity was assessed with I 2 statistics. RESULTS: We included 563 patients from 5 RCTs, of which 251 (42.9%) received spironolactone. Objective assessment of acne improvement (OR 6.59; 95% 3.50-12.43; p < 0.00001; I 2 = 0%) was sixfold higher in the spironolactone group compared with placebo. Subjective assessment showed no difference between the two groups (OR 5.22; 95% 0.62-44.24; p < 0.13; I 2 = 85%). Menstrual irregularities (OR 1.09; 95% 0.37-3.25; p = 0.88; I 2 = 33%) and breast enlargement (OR 1.37; 95% 0.79-2.38; p = 0.26; I 2 = 0%) were nonsignificant in patients taking spironolactone. Trial sequential analysis (TSA) confirmed that the required sample size was reached, favoring spironolactone over placebo. CONCLUSION: Our study suggests that oral spironolactone improves acne in female patients compared to placebo without increasing risks; thus, it should be elevated from "off-label" use to an officially recommended standard of care. PROSPERO REGISTRATION: CRD42024626984.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12359290/

Record 197

pmcid: PMC12547282

pmid: 41140354

doi: 10.1016/j.jdin.2025.08.009

title: Efficacy and safety of twice- or thrice-weekly dutasteride versus daily finasteride in men with androgenetic alopecia: A randomized, investigator-blinded, active-controlled, parallel-group pilot study.

authors

- Sereepanpanich V
- Khunkhet S
- Rojhirunsakool S
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journal: JAAD international

journalAbbr: JAAD Int

year: 2025

pubdate: 2025 Dec

volume: 23

issue:

pages: 69-76

abstract: BACKGROUND: Dutasteride is sometimes used in treating male androgenetic alopecia; however, published data on intermittent regimens are scant. OBJECTIVE: To evaluate efficacy and safety of twice-weekly and thrice-weekly regimens of dutasteride, and to compare with finasteride, in men with androgenetic alopecia. METHODS: Sixty men, aged 21 to 60 years, were randomized to receive dutasteride 0.5 mg twice weekly, dutasteride 0.5 mg thrice weekly, or finasteride 1 mg daily for 24 weeks. Efficacy was evaluated by changes of hair density and diameter using videodermoscopy, and global photographic assessment. RESULTS: Hair density and diameter significantly increased after treatment in 3 groups (all $P < .05$), showing a dose-dependent manner among dutasteride groups. Mean changes of terminal hair count from baseline in twice-weekly dutasteride, thrice-weekly dutasteride, and once-daily finasteride were 7.74, 17.43, and 12.81 hairs/cm ^{2} , respectively. Thrice-weekly dutasteride showed a greater moderate-to-marked improvement than once-daily finasteride (35% vs 21%). Sexual adverse events were similar among 3 groups. LIMITATIONS: Unblinded participants to treatment allocation. CONCLUSION: Twice-weekly and thrice-weekly regimens of dutasteride increased hair growth and were well-tolerated. Although thrice-weekly dutasteride was significantly superior to twice-weekly dutasteride, in terms of hair density and diameter, thrice-weekly dutasteride did not significantly increase hair count over finasteride 1 mg daily.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12547282/

Record 198

pmcid: PMC12021802

pmid: 40292416

doi: 10.3389/fsurg.2025.1562190

title: Efficacy comparison of subcutaneous mastectomy using gasless and gas-insufflation single-port transaxillary approaches for gynecomastia.

authors

- Zhang Y
- Wang H
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- Liu R
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journal: Frontiers in surgery

journalAbbr: Front Surg

year: 2025

pubdate: 2025

volume: 12

issue:

pages: 1562190

abstract: BACKGROUND: To evaluate the clinical efficacy of gasless and single-port gas-insufflation transaxillary approaches in subcutaneous mastectomy for treating patients with gynecomastia (GM). METHODS: This study enrolled 46 patients with GM from May 2022 to October 2023. Twenty patients underwent subcutaneous mastectomy using the single-port gas-insufflation transaxillary approach (gas-insufflation group), while the other 26 patients received the same procedure through the gasless transaxillary approach (gasless group). This study further conducted inter-group comparisons in terms of the operation time, intraoperative bleeding, volume of postoperative drainage, timing of drainage tube removal, short-term postoperative complications, length of postoperative hospital stay, and medical costs. RESULTS: All the 46 patients completed the operation successfully without conversion to open surgery, with confirmed diagnosis of GM through pathology. The average surgical time for the gasless group was significantly shorter than that of the gas-insufflation group (38.20 ± 10.773 vs. 62.96 ± 15.311 min, $P < 0.01$). There were no significant differences between groups in incision length, intraoperative bleeding, unilateral postoperative drainage volume, drainage tube retention time, length of postoperative hospital stay, or postoperative cosmetic outcomes (all $P > 0.05$). CONCLUSION: This study supports the clinical feasibility of using gasless transaxillary approach for subcutaneous mastectomy of patients with GM.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12021802/

Record 199

pmcid: PMC11905218

pmid: 40083863

doi: 10.3892/mco.2025.2831

title: Endocrine paraneoplastic syndromes in lung cancer: A call for clinical vigilance (Review).

authors

- Koloutsou ME
- Soura M
- Andreikos D
- Spandidos DA
- Yavropoulou MP
- Georgakopoulou VE

journal: Molecular and clinical oncology

journalAbbr: Mol Clin Oncol

year: 2025

pubdate: 2025 Apr

volume: 22

issue: 4

pages: 36

abstract: Endocrine paraneoplastic syndromes (PNS) are uncommon but clinically impactful manifestations associated with malignancies, particularly lung cancer, most notably small-cell lung cancer. These syndromes arise from ectopic hormone production by malignant cells, causing systemic effects that are independent of direct tumor invasion or metastasis. Their presentation often precedes or signals recurrence of the primary cancer, underscoring the importance of early recognition and targeted intervention. Common endocrine PNS in lung cancer include syndrome of inappropriate antidiuretic hormone secretion, hypercalcemia, Cushing syndrome, carcinoid syndrome, and other hormonal imbalances such as non-islet cell tumor hypoglycemia, gynecomastia and acromegaly. The pathophysiology of these conditions involves ectopic secretion of bioactive substances such as hormones and cytokines, leading to diverse clinical manifestations. Accurate diagnosis necessitates a combination of biochemical assessments, imaging modalities, and histopathological evaluations to differentiate paraneoplastic processes from primary endocrine disorders. Management strategies emphasize treating the underlying malignancy, often through chemotherapy, radiotherapy, or surgical intervention, alongside symptomatic therapies tailored to the specific endocrine abnormality. Multidisciplinary care is critical for optimizing outcomes and enhancing patients' quality of life. The current review highlights the need for heightened clinical vigilance and a systematic approach to diagnosing and managing endocrine PNS in lung cancer. By fostering early detection and comprehensive management, clinicians can significantly improve prognostic outcomes for affected individuals.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11905218/

Record 200

pmcid: PMC12517656

pmid: 41090008

doi: 10.4103/jfmpc.jfmpc_1858_24

title: Evaluation of oral health during pregnancy and implementation of maternal oral screening tool and referral process in pregnant women: A cross-sectional study.

authors

- Mahajan A
- Singh S
- Upadhyay M
- Jha A
- Pawar M
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journal: Journal of family medicine and primary care

journalAbbr: J Family Med Prim Care

year: 2025

pubdate: 2025 Sep

volume: 14

issue: 9

pages: 3951-3955

abstract: BACKGROUND: A pregnant woman undergoes significant changes in the hormonal levels within her body that can be reflected both systematically as gestational diabetes and as intraoral conditions like gingivitis, dental caries, or pregnancy epulis. The present study aimed to evaluate the oral health, during pregnancy and implementation of maternal oral screening tool and referral process in pregnant women. MATERIALS AND METHODS: This multicenter, cross-sectional prospective study was conducted in Gynaecology outpatient departments after obtaining Ethical clearance from concerned Ethics Committee. Total sample size was 100

pregnant women selected randomly. Inclusion criteria for conducting the study were a) Primigravidae women and b) those, who had given written informed consent. However, exclusion criteria of the study were a) Medications' history, b) use of systemic corticosteroids, c) history of congenital heart diseases, hypertension and diabetes before pregnancy, d) history of chronic renal disease, epilepsy or asthma, e) multiple pregnancies and f) still-born infants. STATISTICAL ANALYSIS: Data analysis was conducted by using the statistical software, SPSS version 20.0 (IBM, NY, USA). Descriptive statistics data was observed as mean, standard deviation (SD) and percentages. Statistical association evaluation was done by "Chi-square" statistical tool. " P " values lower than 0.05 were statistically significant. RESULTS: The preventive oral/dental care was needed in 1.05 ± 0.21, pit and fissure sealants were required in 0.29 ± 0.02, restorations were required in 1.98 ± 0.45, Crown was required in 0.03 ± 0.12, Endodontic treatment was needed in 0.29 ± 0.45 and Tooth extractions were indicated in 0.34 ± 0.12 pregnant female study participants. 56% had gingivitis, 34% were suffering from periodontitis, 7% were diagnosed with dental caries and 3% were diagnosed as having pregnancy epulis. CONCLUSION: It is important to maintain good oral hygiene practices during pregnancy as hormonal levels result in the changes in oral tissues. This study highlights the role of oral screening in pregnant females.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12517656/

Record 201

pmcid: PMC11749746

pmid: 39833036

doi: 10.1097/MD.0000000000041299

title: Exclusive liposuction with glandular tissue redistribution for severe gynecomastia: A case report.

authors

- Yan B
- Hong X
- Hao D
- Zhang J
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- Sun L
- Cang Z
- Chen Y
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journal: Medicine

journalAbbr: Medicine (Baltimore)

year: 2025

pubdate: 2025 Jan 17

volume: 104

issue: 3

pages: e41299

abstract: RATIONALE: Gynecomastia, characterized by abnormal enlargement of male breast tissue, can lead to significant psychological distress, particularly among younger men. Traditional surgical options, such as subcutaneous mastectomy and liposuction, often result in visible scarring and contour deformities. This study introduces the "Stab Flatten" technique, a novel, minimally invasive approach for treating severe gynecomastia, designed to preserve chest aesthetics while minimizing postoperative complications, including scarring and contour irregularities. PATIENT CONCERNS: A 28-year-old male with a 15-year history of progressive bilateral

breast enlargement presented with psychological distress and concerns about the cosmetic outcomes of traditional surgery. He sought a minimally invasive procedure to address both physical discomfort and aesthetic concerns, aiming for minimal scarring. DIAGNOSES: The patient was diagnosed with grade III gynecomastia according to Simon classification, characterized by significant glandular hypertrophy and excess skin. Diagnostic imaging and laboratory tests confirmed the absence of underlying medical conditions. INTERVENTIONS: The patient underwent the "Stab Flatten" technique, which involved exclusive liposuction with glandular tissue redistribution. A 4-mm incision was made at the inframammary crease, and power-assisted liposuction was used to flatten fibroglandular tissue without excision. Postoperative care included a compression garment for 6 months. OUTCOMES: At the 6-month follow-up, the patient showed excellent cosmetic results, with significant improvement in breast contour and symmetry. There were no complications such as hematoma, seroma, or nipple-areola complex deformities. The patient reported high satisfaction with the aesthetic outcome, alongside improved psychological well-being and physical comfort. LIMITATIONS: This study is limited by its single-patient case report, which restricts the generalizability of the findings. Long-term outcomes of the "Stab Flatten" technique require further validation through larger, prospective studies with more diverse patient populations. Additionally, the technique may not be applicable to all grades of gynecomastia or patients with more complex conditions. LESSONS: The "Stab Flatten" technique offers an effective, minimally invasive alternative for treating severe gynecomastia, providing excellent cosmetic and functional outcomes while minimizing scarring. This method may enhance patient satisfaction and reduce recovery times compared to traditional surgical approaches. However, further studies with larger cohorts are necessary to validate its efficacy and generalizability.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11749746/

Record 202

pmcid: PMC12576484

pmid: 40855717

doi: 10.4274/balkanmedj.galenos.2025.2025-8-9

title: Extranodal Rosai-Dorfman Disease of the Breast Presenting as Inflammatory Breast Cancer.

authors

- Bouhani M
- Kamoun S

journal: Balkan medical journal

journalAbbr: Balkan Med J

year: 2025

pubdate: 2025 Oct 31

volume: 42

issue: 6

pages: 565-566

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12576484/

Record 203

pmcid: PMC12551082

pmid: 41133658

doi: 10.3390/epidemiologia6040064

title: Feminizing Adrenocortical Carcinoma in Men: A Rare Cause of Persistent Gynecomastia and a Contemporary Literature Review.

authors

- Arnautu AM
- Paun DL
- Neamtu C
- Gingu C
- Nimigean V
- Tilici DM
- Costinescu R
- Costea M
- Onofrei A
- Grecu B
- Nacea-Radu C
- Paun S

journal: Epidemiologia (Basel, Switzerland)

journalAbbr: Epidemiologia (Basel)

year: 2025

pubdate: 2025 Oct 15

volume: 6

issue: 4

pages:

abstract: BACKGROUND: Feminizing adrenocortical tumors (FATs) are an exceedingly rare subset of adrenal neoplasms, typically affecting adult men and characterized by an excess of estrogen, suppressed gonadotropins, and gynecomastia. Most FATs are malignant, with a poor prognosis and a high risk of recurrence. CASE PRESENTATION: We report the case of a 24-year-old male with bilateral gynecomastia, abdominal mass symptoms, and one year of unexplained infertility. A hormonal evaluation revealed elevated estradiol (90.1 pg/mL) and suppressed ACTH (2.6 pg/mL), with inappropriately normal cortisol levels (12.1 µg/dL). Imaging identified a right adrenal mass. The patient underwent open adrenalectomy, and histopathology confirmed stage II adrenocortical carcinoma (T2NxM0) with autonomous estradiol secretion, negative margins, and a Ki-67 index of 10%. Postoperatively, gonadal function normalized, and infertility resolved at two months. The multidisciplinary tumor board considered but did not initiate adjuvant mitotane, given the completely resected low-stage disease. CONCLUSIONS: This case illustrates the rare presentation of feminizing adrenocortical carcinoma with reversible infertility and highlights the importance of early recognition and close surveillance. In addition, our literature review of 12 male cases reported between 2015 and 2025 emphasizes gynecomastia as the hallmark presentation and discusses emerging evidence supporting active surveillance as a potential alternative to adjuvant mitotane in selected low-risk patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12551082/

Record 204

pmcid: PMC12525205

pmid: 41095859

doi: 10.3390/jcm14196782

title: Fetuin-A Concentration in the Perinatal Period and Maternal BMI Dynamics During Pregnancy, Labor, and Early Postpartum: Is ΔBMI a Parameter Worth Considering?

authors

- Obuchowska-Standyło A

- Kimber-Trojnar Ż
- Czuba M
- Trojnar K
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journal: Journal of clinical medicine

journalAbbr: J Clin Med

year: 2025

pubdate: 2025 Sep 25

volume: 14

issue: 19

pages:

abstract: Background/Objectives : Fetuin-A is a multifunctional glycoprotein involved in metabolic and inflammatory regulation. Although its role in insulin resistance, type 2 diabetes, and cardiovascular disease is well recognized, its relationship with pregnancy-related body mass changes remains unclear. This study aimed to explore associations between maternal BMI dynamics during and shortly after pregnancy and serum fetuin-A concentrations. Methods: Fifty-five healthy Caucasian women with term singleton pregnancies were enrolled. BMI was recorded at three time points: pre-pregnancy, before delivery, and 48 h postpartum. Based on ΔBMI (postpartum minus pre-pregnancy BMI), participants were divided into two groups: ΔBMI ≤ 1 kg/m 2 (n = 32) and ΔBMI > 1 kg/m 2 (n = 23). Serum fetuin-A levels were measured before delivery and postpartum using ELISA. Additional laboratory parameters and body composition were assessed postpartum via standard tests and bioelectrical impedance analysis (BIA). Results: No significant differences were found between groups in BMI at any single time point or in laboratory or BIA-derived parameters. However, all three BMI change indices (ΔBMI_gestational, ΔBMI_puerperal, and ΔBMI) differed significantly between groups. Fetuin-A concentrations did not differ significantly between groups. Importantly, fetuin-A levels decreased significantly after delivery in both groups, suggesting a potential role of the placenta in its regulation. A significant correlation was observed between pre-delivery fetuin-A and postpartum uric acid in Group ΔBMI > 1 kg/m 2 (p = 0.016), indicating a possible link in women with greater gestational weight gain. Conclusions: While fetuin-A was not directly associated with BMI changes, its peripartum dynamics and correlation with uric acid may reflect underlying metabolic-inflammation pathways. ΔBMI indices may offer a more individualized measure of weight dynamics in pregnancy research.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12525205/

Record 205

pmcid: PMC12342181

pmid: 40800655

doi: 10.1093/jscr/rjaf593

title: Galactocele in a prepubertal girl: case report.

authors

- Qarouach S
- Ochan M
- Nassiri H
- Bouljrouf J
- Kisra M

journal: Journal of surgical case reports

journalAbbr: J Surg Case Rep

year: 2025

pubdate: 2025 Aug

volume: 2025

issue: 8

pages: rjaf593

abstract: Galactocele is a rare, benign cystic breast lesion that usually occurs in breastfeeding women, less frequently in children, and more frequently in boys. We report a rare galactocele case in a 2-year-old girl with a painless unilateral breast mass. Imaging was consistent with a cystic lesion, and surgical excision confirmed the diagnosis. Galactocele in this case runs contrary to the male predominance reported in the literature. We discuss the importance of galactocele in the differential diagnosis of breast masses and emphasize its management with monitoring, aspiration, or surgical excision. The etiology remains unknown although it seems to be multifactorial.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12342181/

Record 206

pmcid: PMC12456090

pmid: 40966189

doi: 10.12659/AJCR.948337

title: Gestational Gigantomastia in Multiparity: Successful Management with Bilateral Reduction and Free Nipple Grafts.

authors

- Almajed E
- Alkhelaif R
- Alrasheedi S
- Alshammari MD

journal: The American journal of case reports

journalAbbr: Am J Case Rep

year: 2025

pubdate: 2025 Sep 18

volume: 26

issue:

pages: e948337

abstract: BACKGROUND Gestational gigantomastia is a rare but benign diffuse enlargement of one or both breasts during pregnancy. This report describes the case of a 36-year-old woman with a history of previous pregnancies who presented with back pain associated with bilateral massive breast enlargement that was managed with breast reduction surgery. CASE REPORT A 36-year-old woman, gravida 4 para 3+1, presented 6 months postpartum with significant bilateral breast enlargement, shoulder numbness, and back pain. She was not breastfeeding, and her symptoms began in the second trimester of her fourth pregnancy. There was no evidence of overlying skin changes. Breast length from the sternal notch to the nipple was measured on the right breast at 50 cm and the left at 52 cm. A multidisciplinary team was involved, and the patient underwent bilateral breast reduction with free nipple grafts. A total of 5.150 kg of right breast tissue was excised, measuring 38.0×32.0×8.0 cm. Similarly, 5.530 kg of left breast tissue was excised, measuring 36.0×30.0×9.0 cm, for a total of 10.68 kg of breast tissue. Histopathological investigation confirmed benign breast tissue with fibrocystic changes, with no signs of malignancy. Her postoperative recovery was uneventful, with complete resolution of musculoskeletal symptoms and satisfactory aesthetic outcomes. CONCLUSIONS Gestational gigantomastia requires a multidisciplinary approach. This case highlights its potential to occur in later pregnancies, even without prior history, and demonstrates that bilateral breast reduction with free nipple grafts can offer effective

symptom relief and favorable cosmetic outcomes. Preoperative counseling and individualized treatment planning are essential to optimize patient outcomes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12456090/

Record 207

pmcid: PMC11705995

pmid: 39776831

doi: 10.1093/jscr/rjae835

title: Gestational gigantomastia with pseudoangiomatous stromal hyperplasia - a case report of rare entities.

authors

- Moore S
- Neblett C
- Appiah K
- Thompson R

journal: Journal of surgical case reports

journalAbbr: J Surg Case Rep

year: 2025

pubdate: 2025 Jan

volume: 2025

issue: 1

pages: rjae835

abstract: Gestational gigantomastia (GG) is a rare and severe clinical complication of pregnancy. It is characterized by dramatic and uncontrolled growth of the breasts, often leading to physical discomfort, psychological distress and significant surgical complications. Its pathophysiology is poorly understood; management options include conservative pharmacological and surgical interventions. Pseudoangiomatous stromal hyperplasia of the breast is a very rare, incidental, and histological diagnosis seen predominantly in women aged 30-40 years old, with the management generally involving surgical excision. The authors herein discuss an unusual case of bilateral GG complicated by pseudoangiomatous stromal hyperplasia in a premenopausal Caribbean woman, which is the second reported case in this population, with the first reported by one of our authors.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11705995/

Record 208

pmcid: PMC12643589

pmid: 41293672

doi: 10.1097/GOX.0000000000007290

title: Gestational Gigantomastia: A Century of Evidence with a New Case.

authors

- Fofung SK
- Malik JH
- Fofung WM
- Wabwire B

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2025

pubdate: 2025 Nov

volume: 13

issue: 11

pages: e7290

abstract: BACKGROUND: Gestational gigantomastia (GG) is an extreme, rapidly progressive breast enlargement that can culminate in ulceration, sepsis, cardiopulmonary compromise, and, when care is delayed, maternal death. Scattered case reports leave management uncertain. This study synthesizes more than a century of GG literature and contextualizes a new, complex case in context, clarifying epidemiology, pathogenesis, and treatment pathways. METHODS: Systematic searches of PubMed/MEDLINE, Embase, Ovid Global Health, Google Scholar, and French-language archives (1900-2025) identified 69 publications describing 76 GG episodes. Dual reviewers applied explicit criteria; extracted demographic, clinical, and outcome data; and assessed quality. These findings were integrated with a life-threatening GG case managed at 27 weeks' gestation. RESULTS: Median presentation was 23 weeks (range 6 wk gestation to 1 wk postpartum). Ulceration (53%), hemorrhage (22%), and infection (13%) were common; 4 maternal deaths (5.1%) occurred due to delayed intervention. One-third of assays showed elevated prolactin. First-line medical therapy (bromocriptine, cabergoline, or norethindrone) achieved partial arrest in approximately 35% of trials; definitive control required surgery in 57% (reduction mammaplasty 29%, mastectomy 28%). Maternal outcomes improved when surgery preceded ulceration or systemic decompensation. Our patient recovered fully after an emergency skin-sparing mastectomy, though preterm labor ensued. CONCLUSIONS: GG remains rare, yet potentially fatal. Early recognition, prompt endocrine modulation, and timely, staged surgery are critical. We provide the largest curated dataset to date, quantify the mortality risk, and propose a stepwise management algorithm to guide multidisciplinary teams confronting this formidable obstetric challenge.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12643589/

Record 209

pmcid: PMC11826868

pmid: 39959003

doi: 10.1177/1753495X231173822

title: Gestational gigantomastia: The sentinel event to systemic lupus erythematosus.

authors

- Mohan V
- Singla A
- Agarwal R

journal: Obstetric medicine

journalAbbr: Obstet Med

year: 2025

pubdate: 2025 Mar

volume: 18

issue: 1

pages: 43-45

abstract: Gestational gigantomastia or gravid macromastia is a rare condition presenting as incapacitating breast enlargement during pregnancy. It is physically and psychologically debilitating for the patient. Fewer than 100 cases have been published to date. The etiology of the condition is unknown, but various theories have been proposed. Here we report a case of a 27 year old woman in her second pregnancy, with an abnormal

increase in the size of breasts. Her breast size increased from 34B to 52D brassiere size. Routine antenatal, hormone, and initial autoimmune investigations were within normal limits. The patient was managed conservatively with breast support and had a spontaneous resolution in the postpartum period. During follow up she was diagnosed with systemic lupus erythematosus, making the association of the two conditions a possibility. Good knowledge about this rare condition is necessary for all doctors, especially obstetricians.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11826868/

Record 210

pmcid: PMC12571350

pmid: 41169824

doi: 10.71480/nmj.v66i3.844

title: Gestational Weight Gain and Associated Factors in a Tertiary Hospital in Southwest Nigeria: A Cohort Study.

authors

- Akadri AA
- Grillo EO
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- Adelowo OO
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journal: Nigerian medical journal : journal of the Nigeria Medical Association

journalAbbr: Niger Med J

year: 2025

pubdate: 2025 May-Jun

volume: 66

issue: 3

pages: 1080-1095

abstract: BACKGROUND: Gestational weight gain (GWG) is an important determinant of maternal and foetal wellbeing. Evidence suggests that there is heterogeneity in GWG across countries and regions based on the influence of several factors. This study was designed to investigate GWG, associated factors, and pregnancy outcome among women who gave birth at Babcock University Teaching Hospital, Nigeria. METHODOLOGY: A prospective cohort study carried out on 220 pregnant women who registered for antenatal care before gestational age of 13 weeks and recruited between June 2022 and May 2024. GWG was categorized into adequate, inadequate or excessive according to the United States Institute of Medicine (IOM) guidelines. Logistic regression analysis was used to investigate predictors of GWG, while Relative Risk was reported for pregnancy outcomes. RESULT: Adequate GWG, inadequate GWG and Excessive GWG were achieved by 29.1%, 40.5% and 30.5% of the women, respectively. The median (95% CI) GWG in all study participants was 9.8 (8.8-10.3) Kg. Older maternal age [OR 2.5(1.3-5.0); P=0.013], multiparity [OR 8.2(1.3-51.8); P=0.025] and middle/upper socioeconomic class [OR 14.6(2.1-99.2); P=0.006] predicted increased likelihood of inadequate GWG while being overweight [OR 0.2(0.1-0.4); P<0.001] and obese [OR 0.06(0.02-0.20); P<0.001] were associated with reduced likelihood of inadequate GWG. None of the variables studied predicted excessive GWG. In terms of pregnancy outcome, women with inadequate GWG had a 30% reduced risk of caesarean delivery compared to those with adequate GWG [RR 0.7(0.6-0.9); P= 0.036]. The mean birth weight of babies born to women with inadequate GWG was significantly lower than that of babies born to women with adequate GWG (T-test = 2.998; P=0.003). CONCLUSION: A substantial proportion of study participants had suboptimal GWG (inadequate and excessive), with only a third meeting the IOM recommendations. There is a need for targeted counselling for older, multiparous, and upper socioeconomic class women on adequate GWG.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12571350/

Record 211

pmcid: PMC12213526

pmid: 40607368

doi: 10.3389/fsurg.2025.1617716

title: Giant juvenile phyllodes tumour: a case report.

authors

- Maciulaitis T
- Rimdeikaite M
- Gudaviciene D
- Jakutis N

journal: Frontiers in surgery

journalAbbr: Front Surg

year: 2025

pubdate: 2025

volume: 12

issue:

pages: 1617716

abstract: BACKGROUND: Phyllodes tumours are rare fibroepithelial neoplasms, accounting for less than 1 percent of all breast malignancies, with most cases occurring in women between 40 and 50 years of age. Their occurrence in the paediatric population is highly uncommon, representing less than 10 percent of all phyllodes tumour cases. Due to overlapping clinical and radiological features, these tumours often pose diagnostic challenges, as they are frequently misdiagnosed as fibroadenomas. In younger patients, additional complexities arise from ongoing breast development and the need to minimize long-term aesthetic and functional impact. METHODS: A case report was conducted detailing the clinical presentation, imaging findings, histopathological evaluation, and surgical management of a benign phyllodes tumour in a paediatric female patient. RESULTS: The patient presented with a rapidly growing breast mass, initially suspected to be a fibroadenoma. Surgical excision was performed, and histopathological examination confirmed a benign phyllodes tumour. There were no postoperative complications or recurrence at follow-up. CONCLUSIONS: This case highlights the diagnostic and therapeutic challenges specific to paediatric phyllodes tumours. Given their rarity and potential for misdiagnosis, surgical excision followed by histological evaluation remains crucial for accurate diagnosis. Although treatment principles are generally aligned with adult protocols, adolescents management must also consider breast development and requires a more nuanced surgical approach. Balancing oncological safety with the preservation of breast contour and function introduces unique complexities in this age group. Reporting such cases contributes to the limited literature on juvenile phyllodes tumours and raises awareness of their distinct clinical considerations.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12213526/

Record 212

pmcid: PMC11759554

pmid: 39868055

doi: 10.1016/j.radcr.2024.12.021

title: Giant triple negative pregnancy-associated breast cancer (PABC) in a young woman: From diagnosis to therapy step by step: A case report.

authors

- Bottalico M
- Boldrini C

journal: Radiology case reports

journalAbbr: Radiol Case Rep

year: 2025

pubdate: 2025 Mar

volume: 20

issue: 3

pages: 1686-1690

abstract: Pregnancy-associated breast cancer (PABC) presents unique challenges. This type of breast cancer is often more aggressive than that diagnosed in nonpregnant women, and its diagnosis is frequently delayed. Several factors contribute to this delay, including the physiological changes that occur during pregnancy, such as breast enlargement, breast tenderness and increased tissue density, which can mask early signs of malignancy. Additionally, a lack of awareness among healthcare providers can lead to symptoms being dismissed as normal pregnancy-related changes, and then being underestimated. Consequently, PABC is often diagnosed at more advanced stages, which can result in a poorer prognosis for the patient. This case study reports a 36-year-old woman in good general health, who received histological diagnosis of breast cancer during her pregnancy after presenting for self-detection of an enormous palpable breast mass. Prior to hospitalization, the patient had been treated with antibiotics in the mistaken belief that it was infectious mastitis; subsequently, considering the huge dimensions of the mass and the presence of lymph node metastases, neoadjuvant chemotherapy was started until induction of labor. Imaging techniques such as ultrasonography (and mammography as an adjunct) and nonenhanced MRI (due to potential toxicity of gadolinium to the fetus) can be used for staging maternal breast disease during pregnancy. A multidisciplinary approach which involves the radiologist, the breast surgeon, the oncologist and the gynecologist is fundamental to ensure the best prognostic outcomes for both the mother and the fetus.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11759554/

Record 213

pmcid: PMC12080725

pmid: 41370105

doi: 10.18597/rcog.4258

title: Gigantomastia following the use of a subdermal contraceptive: a case report.

authors

- Montenegro ÁA
- Tamayo Escobar LM
- Sanmiguel Ávila LM
- Kafury Goeta PA
- Reina Ramírez F
- Serna JS

journal: Revista colombiana de obstetricia y ginecologia

journalAbbr: Rev Colomb Obstet Ginecol

year: 2025

pubdate: 2025 Apr 11

volume: 76

issue: 1

pages:

abstract: OBJECTIVE: To describe a case of secondary gigantomastia following the use of a subdermal contraceptive implant, as well as to evaluate the probable causality of the implant as the cause of this condition. MATERIALS AND METHODS: We present the case of a 23-year-old nulligravid patient who, one month after the insertion of a subdermal contraceptive implant, began to experience rapid breast enlargement, mastalgia, localized pruritus, back pain, and an anxiety disorder. Complementary studies ruled out a hormonal, oncological, or pregnancy-related etiology. Two months after the insertion of the subdermal implant, it was removed; however, breast gland enlargement continued after its removal. Reduction mammoplasty was required for definitive treatment. The Naranjo algorithm was applied to determine the likelihood of a causal relationship. CONCLUSIONS: To the best of the authors' knowledge, the probable association between gigantomastia and the use of subdermal contraceptives has not been previously described in the literature. It is important to monitor patients with progesterone implants for the development of gigantomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12080725/

Record 214

pmcid: PMC11666347

pmid: 39587785

doi: 10.1002/npr2.12497

title: Guideline for pharmacological treatment of schizophrenia 2022.

authors

journal: Neuropsychopharmacology reports

journalAbbr: Neuropsychopharmacol Rep

year: 2025

pubdate: 2025 Mar

volume: 45

issue: 1

pages: e12497

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11666347/

Record 215

pmcid: PMC12063920

pmid: 40351746

doi: 10.4103/jmau.jmau_142_20

title: Hamartoma of the Breast in a Young Female: Radio-Pathological Correlation.

authors

- Reddy R

journal: Journal of microscopy and ultrastructure

journalAbbr: J Microsc Ultrastruct

year: 2025

pubdate: 2025 Jan-Mar

volume: 13

issue: 1

pages: 52-53

abstract: Hamartoma of the breast is a rare benign lesion that leads to unilateral breast enlargement with evidence of a localized palpable mass. Ultrasonography findings are typical and include a well-defined mass lesion of heterogeneous echotexture consisting of mixed echogenic and sonolucent areas. This case report describes a hamartoma of the breast in a 17-year-old female.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12063920/

Record 216

pmcid: PMC12120363

pmid: 40367512

doi: 10.2196/70733

title: Identification of Online Health Information Using Large Pretrained Language Models: Mixed Methods Study.

authors

- Tan D
- Huang Y
- Liu M
- Li Z
- Wu X
- Huang C

journal: Journal of medical Internet research

journalAbbr: J Med Internet Res

year: 2025

pubdate: 2025 May 14

volume: 27

issue:

pages: e70733

abstract: BACKGROUND: Online health information is widely available, but a substantial portion of it is inaccurate or misleading, including exaggerated, incomplete, or unverified claims. Such misinformation can significantly influence public health decisions and pose serious challenges to health care systems. With advances in artificial intelligence and natural language processing, pretrained large language models (LLMs) have shown promise in identifying and distinguishing misleading health information, although their effectiveness in this area remains underexplored. OBJECTIVE: This study aimed to evaluate the performance of 4 mainstream LLMs (ChatGPT-3.5, ChatGPT-4, Ernie Bot, and iFLYTEK Spark) in the identification of online health information, providing empirical evidence for their practical application in this field. METHODS: Web scraping was used to collect data from rumor-refuting websites, resulting in 2708 samples of online health information, including both true and false claims. The 4 LLMs' application programming interfaces were used for authenticity verification, with expert results as benchmarks. Model performance was evaluated using semantic similarity, accuracy, recall, F 1 -score, content analysis, and credibility. RESULTS: This study found that the 4 models performed well in identifying online health information. Among them, ChatGPT-4 achieved the highest accuracy at 87.27%, followed by Ernie Bot at 87.25%, iFLYTEK Spark at 87%, and ChatGPT-3.5 at 81.82%. Furthermore, text length and semantic similarity analysis showed that Ernie Bot had the highest similarity to expert texts, whereas ChatGPT-4 showed good overall consistency in its explanations. In addition, the credibility assessment results indicated that ChatGPT-4 provided the most reliable evaluations. Further analysis suggested that the highest misjudgment probabilities with respect to the LLMs occurred within the topics of food and maternal-infant nutrition management and nutritional science and food controversies. Overall, the research suggests that LLMs have potential in online health information identification; however, their understanding of

certain specialized health topics may require further improvement. CONCLUSIONS: The results demonstrate that, while these models show potential in providing assistance, their performance varies significantly in terms of accuracy, semantic understanding, and cultural adaptability. The principal findings highlight the models' ability to generate accessible and context-aware explanations; however, they fall short in areas requiring specialized medical knowledge or updated data, particularly for emerging health issues and context-sensitive scenarios. Significant discrepancies were observed in the models' ability to distinguish scientifically verified knowledge from popular misconceptions and in their stability when processing complex linguistic and cultural contexts. These challenges reveal the importance of refining training methodologies to improve the models' reliability and adaptability. Future research should focus on enhancing the models' capability to manage nuanced health topics and diverse cultural and linguistic nuances, thereby facilitating their broader adoption as reliable tools for online health information identification.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12120363/

Record 217

pmcid: PMC12256661

pmid: 40661323

doi: 10.1002/ccr3.70617

title: IgG4-Related Disease Mimicking Metastatic Malignancy: Polyserositis and Abdominal Lymphadenopathy.

authors

- Huo M
- Li J
- Wu S
- Yan S

journal: Clinical case reports

journalAbbr: Clin Case Rep

year: 2025

pubdate: 2025 Jul

volume: 13

issue: 7

pages: e70617

abstract: In patients presenting with multiple space-occupying lesions, preserved general condition, and normal or mildly elevated tumor markers, IgG4-related disease (IgG4-RD) should be considered in the differential diagnosis. We report an extremely unusual case of IgG4-RD mimicking metastatic malignancy, with polyserositis and widespread abdominal lymphadenopathy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12256661/

Record 218

pmcid: PMC12403221

pmid: 40304397

doi: 10.1093/jbi/wbaf006

title: Imaging Evaluation and Management of Breast Symptoms in the Pediatric Population.

authors

- Johnson AS
- Chapman T

- Tao TY
- Fowler AM

journal: Journal of breast imaging

journalAbbr: J Breast Imaging

year: 2025

pubdate: 2025 Sep 2

volume: 7

issue: 4

pages: 492-506

abstract: The clinical approach, differential diagnosis, diagnostic imaging, and management of breast masses differs between adult and pediatric patients. Breast symptoms in pediatric populations are likely to be normal, variants of normal development, or classically benign. When evaluating pediatric patients, a thorough history and physical examination is key and may include US imaging if clinically indicated. Currently, there are no consensus guidelines from professional societies specific to the imaging evaluation, reporting, or management of the pediatric breast. Inappropriate utilization of additional imaging or biopsy recommendations can cause stress to patients and caregivers, added financial costs, and potential damage to the developing breast. As such, it is increasingly important for radiologists to have a clear understanding of the expected physical and imaging findings for developing breast tissue, developmental abnormalities, benign conditions, and rare malignancies. This review summarizes the expected normal developmental breast changes in the pediatric population as well as common anatomic variants. Both benign and rare malignant breast pathologies are reviewed with a discussion about clinical presentation and management to guide breast imaging trainees and practicing radiologists.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12403221/

Record 219

pmcid: PMC11822279

pmid: 39958496

doi: 10.3348/jksr.2024.0059

title: Imaging Features of the Mesenchymal Tumors of the Breast according to WHO Classification: A Pictorial Essay.

authors

- Lee YJ
- Chang YW
- Lee EJ

journal: Journal of the Korean Society of Radiology

journalAbbr: J Korean Soc Radiol

year: 2025

pubdate: 2025 Jan

volume: 86

issue: 1

pages: 68-82

abstract: Mesenchymal tumors of the breast, which originate from the mammary stroma, are rare accounting for only approximately 0.5%-1% of all breast tumors. Pathologically, they can exist on a spectrum, ranging from benign to malignant. Such tumors may present with nonspecific findings on breast imaging, including mammography, ultrasound, and MRI, which can lead to diagnostic challenges. In the 2019 revised 5th edition of

the World Health Organization classification, breast mesenchymal tumors are categorized into six groups. The current pictorial essay aimed to explore the clinical, pathological, and imaging characteristics of representative lesions in each category according to this six-group classification, with the ultimate goal of enhancing awareness for early diagnosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11822279/

Record 220

pmcid: PMC12204885

pmid: 40038218

doi: 10.1007/s11604-025-01763-z

title: Imaging insights of FDG-PET from neonates to infants.

authors

- Minamimoto R
- Abe Y
- Kamiya S
- Nakane T
- Ito R
- Kato K
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journal: Japanese journal of radiology

journalAbbr: Jpn J Radiol

year: 2025

pubdate: 2025 Jul

volume: 43

issue: 7

pages: 1066-1077

abstract: In pediatric oncology, 18 F-fluorodeoxyglucose (FDG)-positron emission tomography (PET)/computed tomography (CT) is valuable as a tool for noninvasive imaging and monitoring. While many reports have reviewed the use of PET and PET/CT in pediatrics, considerable variations in age, body size, and metabolism are seen during different stages of childhood development. Neonates (from birth to one month old) and infants (from 1 month to 1 year) present unique challenges for FDG-PET/CT examination due to their small body size, the immaturity of organs, the need for specialized patient preparation, and support requirements during scanning. In addition, differences in metabolic activity can lead to distinct differences in patterns of physiological FDG uptake on PET/CT imaging between neonates and infants. These factors differ significantly from those encountered in older children, who may be treated similarly to adults during imaging procedures. This review, based on both the literature and clinical experience, explores the specific characteristics, challenges, and considerations for FDG-PET/CT imaging from neonates to infants, with a focus on optimizing imaging protocols and interpreting physiological variations in this growth period.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12204885/

Record 221

pmcid: PMC12178523

pmid: 40547715

doi: 10.4103/jfmpc.jfmpc_1052_23

title: Impact on maternal mental health on infants development: A narrative review.

authors

- Devgade P
- Patil M

journal: Journal of family medicine and primary care

journalAbbr: J Family Med Prim Care

year: 2025

pubdate: 2025 May

volume: 14

issue: 5

pages: 1584-1588

abstract: Mental health problems among expectant mothers and new mothers are important yet neglected health problems that significantly raise maternal and child morbidity and death in India. The mental health and well-being of parents have an impact on the growth and development of their children. Maternal depression during pregnancy and after delivery reduces the chance that a child survives since it signals that the baby will grow slowly and have a higher chance of developing diarrhea. More specifically, depression, one of the most common causes of disease-related limitation in women, not only impacts the women but also has an adverse effect on their children. This may impair a mother's capacity to care for her children, increasing the possibility that the kids will suffer from sickness, malnutrition, delayed development, and mental problems that could linger into adulthood. Reduced child growth has adverse effects on short-term and long-term effects on non-communicable illness risks, learning capacity, working ability, academic achievement, brain development, and non-communicable disease risks.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12178523/

Record 222

pmcid: PMC12209325

pmid: 40599418

doi: 10.1002/hsr2.70961

title: Investigating the Characteristics of Pelvic Ultrasound in Girls With Their First Menstrual Cycle: A Two-Center Cross-Sectional Study.

authors

- Rashed MM
- Eshraghi P
- Hassannejad E
- Jafarpour M

journal: Health science reports

journalAbbr: Health Sci Rep

year: 2025

pubdate: 2025 Jul

volume: 8

issue: 7

pages: e70961

abstract: BACKGROUND AND AIMS: One of the important modalities in the case of precocious puberty diagnosis is pelvic ultrasonography. This method provides information about the internal uterus and ovaries. The aim of this study is to assess pelvic sonography parameters in Iranian girls who experience menarche.

METHOD: This cross-sectional study was conducted in the radiology department of Akbar and Dr. Sheikh hospitals in 2022. The onset of puberty symptoms should not have been before 8 years old. A pediatric endocrinologist visited all the participants to ensure a normal puberty process. An expert radiologist conducted ultrasound parameters measurements. A predesigned questionnaire was used to assess the demographic and anthropometric data, and all data were analyzed. RESULT: Fifty girls were enrolled in the study with a median age of 11 years old. The mean of the longitudinal diameter of the uterus was 61.8 ± 7.59 mm, the mean of the transverse diameter of the uterus was 35.91 ± 5.01 mm, the mean of the anteroposterior diameter of the uterus was 24.76 ± 4.81 mm, and the mean of the uterus volume was 30.04 ± 11.84 cm^3. The association of weight, height, and BMI with different sonography measurements was assessed. No significant association between these findings and BMI was found ($p > 0.05$); however, a significant correlation was found between height and anteroposterior diameter of the uterus ($p = 0.015$). Linear regression analysis showed a positive relationship between height and anteroposterior diameter of the uterus ($p = 0.027$). CONCLUSION: The ultrasonography measurements may differ among different populations, and studies should be conducted on different ethnicities. Our results were comparable to those of other studies; however, the age differed, and the studies did not focus on menarche time.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12209325/

Record 223

pmcid: PMC11699747

pmid: 39672031

doi: 10.1016/j.ijscr.2024.110686

title: Juvenile breast hypertrophy: Successful breast reduction with a loss of 16 % of body weight, with no recurrence observed at 2-year follow-up, a rare case report.

authors

- Benali S
- Benlghazi A
- Essaoudi MA
- Imane Z
- Elhassani MM
- Kouach J

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2025

pubdate: 2025 Jan

volume: 126

issue:

pages: 110686

abstract: INTRODUCTION AND IMPORTANCE: Juvenile breast hypertrophy, also known as juvenile macromastia or juvenile gigantomastia, is a rare disorder characterized by rapid, excessive breast growth in prepubertal or peripubertal girls, with no apparent cause. Juvenile breast hypertrophy is considered one of the most challenging mastopathies to manage. CASE PRESENTATION: We present the case of a 12-year-old Moroccan girl with virginal breast hypertrophy, who was successfully treated. CLINICAL DISCUSSION: Juvenile breast hypertrophy is a benign disorder characterized by breast enlargement without the presence of masses. It can present diagnostic challenges for physicians and necessitates a multidisciplinary approach for accurate diagnosis, supported by histological findings of stromal and ductal hyperplasia. Additionally, it may lead to significant physical and psychological effects on young girls. Surgery is regarded as the preferred treatment and may involve either breast reduction or bilateral mastectomy. This approach is warranted due to the size of the breasts and their associated physical and psychological impacts, as experienced by our patient.

CONCLUSIONS: This report aims to share knowledge about this rare case and may also pave the way for further research to understand the underlying mechanisms better and standardize the treatment of juvenile breast hypertrophy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11699747/

Record 224

pmcid: PMC12414121

pmid: 40923046

doi: 10.1177/11782234241306209

title: Juvenile Fibroadenoma Detected at 2 Years and Rapid Growth at Thelarche: Case Report and Literature Review.

authors

- Schaefgen B
- Buchen S
- Sinn HP

journal: Breast cancer : basic and clinical research

journalAbbr: Breast Cancer (Auckl)

year: 2025

pubdate: 2025

volume: 19

issue:

pages: 11782234241306209

abstract: Juvenile fibroadenoma typically is seen in the adolescent patient and has distinctive clinical and histopathological features that are related to the early onset of the benign tumor. Only rarely can juvenile fibroadenoma occur as a prepubertal lesion or may be detected earlier in childhood, raising questions about differential diagnosis and management of the lesion. We present a case of very early first manifestation of juvenile fibroadenoma in a 2-year-old patient. Under clinical observation and conservative management over a 7-year period, there was no progression. At the age of 9 years, rapid growth of the lesion occurred causing clinical symptoms. Complete surgical excision resulted in a good clinical outcome. Histologically, the diagnosis of juvenile fibroadenoma was made. In summary, juvenile fibroadenoma is the most common breast neoplasia in adolescents and may cause excessive unilateral growth resulting in gross asymmetry and pain. However, it can be detectable in early childhood and after a dormant period of several years, as in this case, lead to sudden massive growth at the onset of puberty. The clinical management should include an interdisciplinary treatment approach with gynecology, pediatrics, and pediatric surgery to balance the risks and benefits of conservative management. Surgical removal can be safely postponed until it becomes necessary due to clinical symptoms.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12414121/

Record 225

pmcid: PMC12123941

pmid: 40435211

doi: 10.1080/20565623.2025.2510871

title: Knowledge and awareness of breast cancer signs and symptoms among Jordanian women.

authors

- Alrosan AZ
- Alwidyan T

- Heilat GB
- Rataan AO
- Madae'en S
- Alrosan K
- Awwad FS
- Ali T

journal: Future science OA

journalAbbr: Future Sci OA

year: 2025

pubdate: 2025 Dec

volume: 11

issue: 1

pages: 2510871

abstract: BACKGROUND: Breast cancer (BC) remains the most common cancer among women worldwide. Early detection plays a vital role in reducing morbidity and mortality. This study aimed to assess Jordanian women's awareness of BC signs and symptoms to support timely recognition and intervention. RESEARCH DESIGN AND METHODS: A nationwide cross-sectional study was conducted involving 381 Jordanian women from the general community. Participants completed a self-administered questionnaire assessing their knowledge of BC signs and symptoms using a three-point Likert scale ("Yes," "Unsure," "No"). Demographic variables were analyzed for association with knowledge levels using Pearson's Chi-square or Fisher's Exact Test. RESULTS: 80.3% of participants demonstrated good awareness of BC signs and symptoms. Significant associations were found between knowledge levels and marital status (p = 0.000), age (p = 0.033), having children (p = 0.000), number of children (p = 0.003), education specialty (p = 0.000), and working area (p = 0.005). CONCLUSIONS: Most participants demonstrated a strong awareness of BC signs and symptoms. However, educational interventions are needed to target knowledge gaps, particularly among unmarried women.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12123941/

Record 226

pmcid: PMC12465561

pmid: 41013463

doi: 10.1186/s12905-025-03994-8

title: Knowledge and awareness of breast cancer symptoms, risk factors, and screening barriers among women in Kuwait: a cross-sectional study.

authors

- AlMutawah K
- Taqi G
- Radhwan S
- AlMutairi A
- AlHussainan A
- Faraj M
- AlBaloul AH

journal: BMC women's health

journalAbbr: BMC Womens Health

year: 2025

pubdate: 2025 Sep 26

volume: 25

issue: 1

pages: 448

abstract: BACKGROUND: Breast cancer is the most diagnosed cancer globally and the second leading cause of cancer-related mortality across all demographics. In Kuwait, the incidence is 50 cases per 100,000 person-years. Delayed detection, often linked to limited awareness of breast cancer symptoms and risk factors, contributes significantly to morbidity and mortality. This study aimed to assess breast cancer knowledge, awareness of symptoms and risk factors, and perceived barriers to screening among women in Kuwait. METHODS: A cross-sectional survey was conducted among 490 employed women aged 18 years and older across Kuwait. A structured questionnaire was used to assess participants' awareness of breast cancer symptoms and risk factors, as well as barriers to screening. Composite scores were developed for symptom awareness, risk factor knowledge, and screening barriers. Multiple linear regression was used to identify demographic predictors associated with each score. RESULTS: The median participant age was 35 years, with 66.1% holding a bachelor's degree. Higher education levels were significantly associated with greater BC sign identification ($p = 0.013$ for bachelor's degree; $p = 0.009$ for postgraduate degree). Participants with a family history of breast cancer demonstrated significantly greater knowledge of both BC signs and risk factors ($p < 0.001$). The most recognised symptom was a breast lump or thickening (72.0%). The most frequently cited barrier to screening was concern about pain or discomfort (57.8%). CONCLUSION: While women in Kuwait have moderate awareness of key breast cancer symptoms and risk factors, knowledge gaps and screening barriers persist, particularly regarding lesser-known warning signs and screening apprehension. Public health strategies should prioritise targeted education initiatives, including workplace wellness programs and culturally tailored awareness campaigns, to improve early detection and reduce the breast cancer burden in the region.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12465561/

Record 227

pmcid: PMC11981309

pmid: 40212224

doi: 10.1097/MS9.0000000000003117

title: Knowledge of breast cancer among nurses in a tertiary care center: a descriptive cross-sectional study.

authors

- Shrestha Simangainda DM
- Barma A
- Rawal G
- Karn M
- Jeevan BK
- Adhikari J

journal: Annals of medicine and surgery (2012)

journalAbbr: Ann Med Surg (Lond)

year: 2025

pubdate: 2025 Apr

volume: 87

issue: 4

pages: 1904-1908

abstract: OBJECTIVE: This study aims to assess the knowledge and awareness of breast cancer among nurses, a critical component of the healthcare workforce. While many previous studies have focused on female

participants and students, this study specifically targets nurses working in a tertiary care center, given their crucial role in patient education, early cancer detection, and facilitating timely medical intervention. METHODS: A descriptive cross-sectional study was conducted among 235 nurses working in the various departments of a tertiary care center from August to September 2021. Data on breast cancer knowledge were collected using structured questionnaires with knowledge levels categorized as "good" or "poor" using the median score as a cut-off. The validity and reliability of the questionnaire were ensured through expert review and pilot testing. Descriptive statistics were used to present the findings. RESULTS: Our study revealed overall good knowledge (64.7% correct answers) about breast cancer among the study participants. Most participants (93.2%) were aged 18-29, and 72.3% held a Proficiency Certificate Level in Nursing (PCL) nursing qualification. Only half of the respondents recognized obesity and smoking as risk factors, with common misconceptions including the belief that nulliparity and alcohol consumption are not risks. While over 90% identified breast lumps and enlargement as symptoms, only half recognized non-lump symptoms like nipple discharge and skin changes. Participants were highly aware of breast cancer detection tools, with nearly all recognizing clinical breast exams, mammography, and breast ultrasound. About 75.7% knew of biopsy, and 74.5% mentioned magnetic resonance imaging. Mammography was the most commonly recognized screening tool (95.7%). CONCLUSION: While general awareness of breast cancer is high among nurses, specific gaps remain regarding certain risk factors and symptoms, including obesity, smoking, and non-lump breast cancer indicators. Targeted education through workshops or continuous medical education programs could enhance nurses' ability to effectively educate and screen patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11981309/

Record 228

pmcid: PMC12102407

pmid: 40397234

doi: 10.1007/s44446-025-00011-9

title: Knowledge, attitude and practice towards anabolic-androgenic steroids utilisation among gymnasium attendees in Al-Qassim Province, Saudi Arabia.

authors

- Alsugoor MH

journal: Saudi pharmaceutical journal : SPJ : the official publication of the Saudi Pharmaceutical Society

journalAbbr: Saudi Pharm J

year: 2025

pubdate: 2025 Apr 24

volume: 33

issue: 1-2

pages: 4

abstract: Despite the serious side effects associated with anabolic-androgenic steroids (AAS) being well-documented, their widespread misuse for bodybuilding and endurance enhancement continues to be a concern worldwide. A substantial number of studies have been conducted, both internationally and in Saudi Arabia, to investigate knowledge, attitudes, and practices with respect to AAS use among gym users. These studies have been conducted in various cities throughout Saudi Arabia and have focused primarily on male gym users; however, no local studies have been conducted in Al-Qassim province. Therefore, this study aimed to investigate the knowledge, attitudes, and practices around AAS use of both male and female gym users in Al-Qassim Province. This community-based, cross-sectional, questionnaire-based study was conducted from January 2022 to March 2022 and recruited 415 gym users from 24 gyms in Al-Qassim. In the results, 62.9% of the participants were men, and 37.1% were women. The participants demonstrated diverse levels of knowledge about AAS: 74% had heard about AAS, and 72.8% and 71.6% were aware of AAS's effects on muscle and body weight, respectively. However, only 13.3% knew that AAS could cause breast enlargement. Regarding participants' attitudes towards AAS, the majority of the participants (83.6%) believed that using AAS harms

health, and 52% and 52.8% of participants believed that there should be punishments for people who use or sell AAS, respectively, while 62.4% of participants felt sorry for AAS users. Our findings on participants' practices showed that 37.3% of participants knew someone who uses AAS, and most importantly, that 19.5% of the gym users (19.2% and 20.1% of male and female users, respectively) admitted that they have used AAS. 51.1% of the participants believed that these AAS are easy to obtain. The study also found a significant relationship between AAS use and sociodemographic variables, including marital status ($p = 0.031$), time spent in the gym ($p = 0.016$), chronic diseases ($p = 0.028$), and smoking ($p = 0.031$). In conclusion, despite the participants' relatively good knowledge, attitudes, and practices concerning AAS use, they lack knowledge on certain side effects. This could direct policymakers in managing and acting to increase awareness for bodybuilders and the public. In addition, the high prevalence of AAS users could alert the ministries of health and other related ministries to the need for strict rules, especially those stopping or at least minimising accessibility to these substances.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12102407/

Record 229

pmcid: PMC12396867

pmid: 40894814

doi: 10.1055/s-0045-1802630

title: Lipomas in Patients of Gynecomastia: Is It of Any Relevance?

authors

- Punia S
- Goel A
- Gupta A

journal: Indian journal of plastic surgery : official publication of the Association of Plastic Surgeons of India

journalAbbr: Indian J Plast Surg

year: 2025

pubdate: 2025 Aug

volume: 58

issue: 4

pages: 325-326

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12396867/

Record 230

pmcid: PMC12447314

pmid: 40978063

doi: 10.3389/fonc.2025.1635515

title: Male accessory breast carcinoma: a rare case report and literature review.

authors

- Luo C
- Wu S
- Zeng Y
- Zhao S

journal: Frontiers in oncology

journalAbbr: Front Oncol

year: 2025

pubdate: 2025

volume: 15

issue:

pages: 1635515

abstract: BACKGROUND: Accessory breast carcinoma, arising from embryologically persistent mammary tissue, is exceptionally rare in males, accounting for 2.4%-5.3% of all accessory breast malignancies. Due to limited clinical data, standardized diagnostic and therapeutic protocols remain undefined. CASE PRESENTATION: A 72-year-old male presented with a 3-year history of a painless, mobile right axillary mass. Ultrasonography revealed a solid hypoechoic nodule with moderate vascularity. Histopathological examination confirmed invasive ductal carcinoma (grade II) with infiltrative growth, ER/PR/AR positivity (90%), and TRPS1 expression, confirming mammary origin. Adjuvant therapies were declined by the patient. DISCUSSION: Differential diagnoses for axillary masses include fibroadenoma, lymphadenopathy, and cutaneous adnexal tumors. Immunohistochemistry (TRPS1, GATA-3) and histomorphology (absence of apocrine differentiation) are pivotal for distinguishing accessory breast carcinoma from mimics. Current management aligns with male breast cancer guidelines, emphasizing surgical resection, lymph node dissection, and adjuvant therapies tailored to molecular profiles. Over 90% of male breast cancers are hormone receptor-positive, warranting endocrine therapy. CONCLUSION: Male accessory breast carcinoma poses significant diagnostic challenges due to its rarity and nonspecific presentation. Clinicians should consider this entity in differential diagnoses of axillary or inguinal masses, irrespective of patient sex. Core needle biopsy and advanced imaging aid preoperative evaluation. Multimodal treatment, including surgery and risk-stratified adjuvant therapies, is critical for optimizing outcomes. This case underscores the need for heightened clinical suspicion and evidence-based guidelines to address this understudied malignancy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12447314/

Record 231

pmcid: PMC12605550

pmid: 40913704

doi: 10.1007/s11547-025-02084-x

title: Male breast MRI: a review of different pathological conditions.

authors

- Barillari M
- Zanutto P
- Pellini F
- Fiorio E
- Deguidi G
- Invento A
- Nottegar A
- D'Onofrio M
- Mansueto G

journal: La Radiologia medica

journalAbbr: Radiol Med

year: 2025

pubdate: 2025 Nov

volume: 130

issue: 11

pages: 1752-1766

abstract: The male breast is predisposed to be affected by many of the same pathological processes as the female breast is. The diagnosis of male breast pathologies is generally achievable when clinical evaluation is combined with standard breast imaging methods such as mammography and ultrasound. Magnetic resonance imaging is also a valuable tool in diagnosing the main pathologies affecting the male breast, especially for evaluating pre- and post-surgical treatments and follow-up. However, although this technique has been sufficiently regulated and adopted by many breast radiologists for female breast imaging, its application in the diagnosis of male breast pathologies remains limited to a few specialized centers. This article, based on a retrospective analysis of the experience of the University of Verona, explores various aspects of male breast diseases, including benign conditions such as gynecomastia and breast implant ruptures in transgender women as well as malignant entities such as male breast cancer. Emphasis is placed on the distinctive morphological features, enhancement patterns and kinetics observed in male breast lesions on dynamic contrast-enhanced MRI. This article provides a comprehensive overview of the application of MRI in male breast disease assessment, highlighting the potential role of MRI as a complementary tool to traditional breast imaging techniques.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12605550/

Record 232

pmcid: PMC12578593

pmid: 39435535

doi: 10.1308/rcsann.2024.0076

title: Mammary myofibroblastoma of the male breast: a case report and literature review.

authors

- Elayyan R
- Rizk M
- Shah C
- Price R
- Garg N

journal: Annals of the Royal College of Surgeons of England

journalAbbr: Ann R Coll Surg Engl

year: 2025

pubdate: 2025 Nov

volume: 107

issue: 8

pages: 601-608

abstract: Mammary myofibroblastoma (MFB) is a rare benign spindle cell tumour predominantly affecting males, but also observed in postmenopausal females. Its diagnosis remains challenging owing to overlapping histological features with malignant lesions and limited tissue sampling in core biopsies. We present a case of incidentally discovered mammary MFB in a 63-year-old man and review its clinical, radiological and histopathological characteristics. The patient, who had a history of distal pancreatectomy and splenectomy, presented with an incidental left anterior chest wall nodule discovered on computed tomography scan. Clinical examination revealed a benign left retroareolar lump, confirmed by breast ultrasound and mammography. Ultrasound-guided core biopsy demonstrated characteristic spindle cells, prompting immunohistochemical staining confirming the diagnosis of MFB. The lesion was surgically excised with clear margins. Mammary MFB is commonly seen in postmenopausal women and older men, presenting as painless, mobile breast lumps.

Imaging findings are nonspecific, resembling fibroadenomas or fat necrosis. Histologically, MFB lacks mammary ducts or lobules and displays characteristic spindle cells with collagenous stroma. Immunohistochemistry aids in differentiating it from other spindle cell tumours. Surgical excision is curative, with no reported cases with recurrence. Mammary MFB should be considered in the differential diagnosis of breast masses in males and postmenopausal women. Despite the challenges in diagnosis, its benign nature and favourable prognosis warrant timely recognition and appropriate management through surgical excision. Further research is needed to establish clear management guidelines and explore its underlying pathogenesis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12578593/

Record 233

pmcid: PMC12318540

pmid: 40755643

doi: 10.7759/cureus.87289

title: Management of a Complex Fibroadenoma During Pregnancy.

authors

- Wills M
- Clary K
- Yellin J
- Krupa K

journal: Cureus

journalAbbr: Cureus

year: 2025

pubdate: 2025 Jul

volume: 17

issue: 7

pages: e87289

abstract: This case report describes a 25-year-old woman who presented with a complex fibroadenoma with multifocal cysts and lactational changes during pregnancy. The fibroadenoma grew rapidly during the patient's pregnancy, necessitating surgical intervention postpartum. While conservative management is generally preferred during pregnancy and lactation, this case highlights the importance of prompt diagnosis, timely surgical intervention, and the management of diagnostic and surgical challenges in pregnancy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12318540/

Record 234

pmcid: PMC12081592

pmid: 40374961

doi: 10.1007/s00383-025-06039-1

title: Management of neonatal ovarian cysts: a 10-year single-center experience.

authors

- Szymon O
- Fryczek M
- Kotlarz A
- Taczanowska-Niemczuk A
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journal: Pediatric surgery international

journalAbbr: Pediatr Surg Int

year: 2025

pubdate: 2025 May 15

volume: 41

issue: 1

pages: 135

abstract: INTRODUCTION: Postnatal management of ovarian cysts in neonates is debated. METHODS: A retrospective analysis of neonates diagnosed with ovarian mass at a tertiary referral center (Jan 2014-May 2024). RESULTS: 64 neonates (1 day-8 months; mean: 8.2 weeks) were diagnosed with 65 ovarian masses ≥ 20 mm (mean: 44.2 mm; range: 20-81 mm). Prematurity was present in 31% (n = 20). Primary surgery was performed in 11 cases due to severe symptoms/uncertain mass origin; 3 underwent percutaneous puncture; 50 were observed. Of these, 37/50 (74%) had cyst regression within 18 months, including 7 complex cysts (19%). Delayed involution occurred in 6/50 (12%) observed. Both complex morphology and larger cyst size were significantly associated with delayed regression (p = 0.0005, p = 0.0045, respectively). 8/50 (16%) underwent intervention for cyst enlargement (2 laparoscopies, 5 punctures) or concern for interval torsion (1 laparotomy). No confirmed postnatal torsion or hemorrhagic complications occurred in the observation group. One patient required a repeat procedure after percutaneous reduction. No significant association was found between lesion size and the symptoms of recent adnexal torsion (p = 0.99). CONCLUSIONS: Observation is safe for asymptomatic ovarian cysts. Postnatal torsion is rare but can occur regardless of lesion size, including patients with no prior history of ovarian cysts.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12081592/

Record 235

pmcid: PMC12682638

pmid: 41367860

doi: 10.3389/fonc.2025.1634166

title: Management of ovarian granulosa cell tumor in childhood: a case report and recommendations for a multidisciplinary approach.

authors

- Arnaboldi SMC
- Gattuso G
- Nicolosi ML
- Molinari S
- Bettini LR
- Leva E
- Passoni P
- Chiappa V
- Daolio C
- Balduzzi AC
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journal: Frontiers in oncology

journalAbbr: Front Oncol

year: 2025

pubdate: 2025

volume: 15

issue:

pages: 1634166

abstract: Juvenile granulosa cell tumor (jGCT) is a rare subtype of pure sex-cord tumors, mostly affecting patients under 20 years of age. As this tumor originates from ovarian cells specialized in sex steroid secretion, jGCT can present with endocrine disorders, including precocious puberty in prepubertal girls or menstrual irregularities in postpubertal patients. In other cases, jGCT may manifest with symptoms related to pelvic mass effects or acute abdomen, prompting urgent gynecologic or surgical evaluation. Most patients are diagnosed with ovarian-confined disease, and for these patients, the survival rates exceed 90% following surgery alone. However, advanced and relapsed disease remains a significant concern. As the survival rates for cancer continue to improve, addressing survivorship care is essential. Long-term follow-up for patients diagnosed with jGCT in childhood requires a multidisciplinary approach. We hereby describe a clinical case of jGCT diagnosed in an infant girl for whom a comprehensive multidisciplinary care plan was arranged. Through a comprehensive review of the literature, we developed a clinically applicable flowchart for the multidisciplinary management of jGCT at diagnosis and during follow-up, emphasizing the need for patient-centered care that integrates the work of oncologists, endocrinologists, surgeons, gynecologists, and geneticists.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12682638/

Record 236

pmcid: PMC12408297

pmid: 40917355

doi: 10.3389/fendo.2025.1632257

title: McCune-Albright syndrome with multiple hyperfunctional endocrinopathies: diagnosis, treatment, and long-term follow-up: a case report.

authors

- Zainab MJ
- Khamies LL
- Baladi J
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journal: Frontiers in endocrinology

journalAbbr: Front Endocrinol (Lausanne)

year: 2025

pubdate: 2025

volume: 16

issue:

pages: 1632257

abstract: McCune-Albright syndrome (MAS) is a rare genetic disorder characterized by a triad of café-au-lait spots, fibrous dysplasia, and hyperfunctioning endocrinopathies, resulting from a mosaic mutation in the guanine nucleotide-binding protein (GNAS) gene. This case report presents the long-term follow-up of an eight-year-old girl diagnosed with MAS, who first presented at 22 months of age with skin pigmentation, hyperthyroidism, and precocious puberty, later developing additional features such as fibrous dysplasia and growth hormone excess. This complex presentation of MAS-featuring more than two hyperfunctioning endocrinopathies along with fibrous dysplasia-has rarely been described in the literature. The patient was

appropriately managed with a combination of carbimazole (an imidazole), letrozole (an aromatase inhibitor), lanreotide (a somatostatin analogue), and zoledronic acid (a bisphosphonate). Notably, this is the first reported use of lanreotide in the management of MAS in a pediatric patient. This case highlights the challenges of managing MAS over an extended period, particularly when multiple endocrinopathies are present from an early age. We describe an effective approach to treatment, emphasizing how each condition was managed while considering the interactions among the various manifestations. Given the rarity of long-term follow-up reports, this case provides valuable insights into the management of MAS in pediatric patients with complex, multisystem presentations.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12408297/

Record 237

pmcid: PMC11806124

pmid: 39786527

doi: 10.1007/s10815-024-03307-2

title: Mechanism of female CHH caused by compound heterozygous mutations in the LHB gene.

authors

- Ma H
- Li C
- Gao J
- Wu W
- Sun Z
- Wang X
- Nie M
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- Mao J
- Han Q

journal: Journal of assisted reproduction and genetics

journalAbbr: J Assist Reprod Genet

year: 2025

pubdate: 2025 Jan

volume: 42

issue: 1

pages: 293-302

abstract: BACKGROUND: Luteinizing hormone (LH) plays a crucial role in the postnatal development and maturation of gonads. Inactivating mutations of the luteinizing hormone beta subunit (LHB)gene are extremely rare and can result in congenital hypogonadotropic hypogonadism (CHH). METHODS: We conducted DNA sequencing on an 18-year-old female patient with undetectable LH and clinical symptoms of CHH. Pulsatile GnRH was administered to promote puberty development. In vitro construction of mutant genes, confocal microscopy, and protein functional assays were used to investigate the effects of genetic variants on hormone function and secretion. Experiments were conducted in HEK293T cells to examine the colocalization and dimerization of LH subunits, as well as to measure intracellular and extracellular LH concentrations. RESULTS: Compound heterozygous mutations of c.252C>G (p.F84L) and c.364G>A (p.G122S) were found in the patient's genome. Pulsatile GnRH therapy was effective in promoting puberty development and ovulation. The LH alpha subunit was found to co-localize with both mutant beta subunits after immunofluorescence staining, and immunoprecipitation detected the dimerization of the LH alpha subunit with both mutant beta subunits. Higher intracellular LH concentrations and lower extracellular LH concentrations compared to the wild type indicate

secretion dysfunction for LH. CONCLUSION: Compound heterozygous mutations of c.252C>G (p.F84L) and c.364G>A (p.G122S) in the LHB gene may lead to CHH for female patient. These mutations do not impair the expression and dimerization of the alpha and beta subunits, but they do prevent the secretion of LH. The study expands our understanding of the clinical manifestation of LHB gene mutations in females and provides a treatment for these patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11806124/

Record 238

pmcid: PMC12334347

pmid: 40786363

doi: 10.7759/cureus.87631

title: Mitochondrial Impairment in Healthy Controls With Recent Headache and Multiple Symptoms.

authors

- Golomb BA
- Situ JY
- Hamilton G

journal: Cureus

journalAbbr: Cureus

year: 2025

pubdate: 2025 Jul

volume: 17

issue: 7

pages: e87631

abstract: BACKGROUND: Persistent headache and multiple symptoms have each been tied to mitochondrial dysfunction. We assess whether reported recent headache and multiple symptoms (last two weeks) relate to bioenergetics in individuals who do not meet criteria for chronic multisymptom illness (CMI). METHODS: Twenty participants screening negative for CMI rated the presence/absence and severity (0-10) of recent (last two weeks) headache and multiple symptoms and underwent 31 P-MRS (phosphorus-31 magnetic resonance spectroscopy) to assess the post-exercise phosphocreatine (PCr) recovery time constant (PCr-R). Phosphocreatine is depleted with exercise, and the post-exercise PCr recovery rate depends on the ATP production rate. RESULTS: Eighteen of 20 participants successfully depressed PCr (on 31 P-MRS) with exercise, enabling PCr-R assessment. Five participants (28%) reported recent headache. Headache presence was related to PCr-R (seconds): PCr-R mean±SD without headache: 34.6±11.5; PCr-R mean±SD with headache 62.3±22.6, p=0.003. Headache severity (0-10) was correlated with PCr-R (seconds): r=0.79, p=0.0001. Headache presence and severity were related to greater multiplicity of recent nonheadache symptoms: headache severity vs summed nonheadache symptom severity, r=0.68, p=0.001 (correlation to summed symptom ratings without excluding headache, r=0.72, p=0.0004). Longer PCr-R predicted greater summed symptom severity: regression β=0.47 (SE=0.15), p=0.007. CONCLUSION: Recent headache and multiple symptoms were related to prolonged post-exercise PCr-recovery in persons screening negative for CMI. Future studies should distinguish among potential foundations for these associations.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12334347/

Record 239

pmcid: PMC12543815

pmid:

doi: 10.1210/jendso/bvaf149.1462

title: MON-135 Pituitary Adenoma Presenting with Gynecomastia.

authors

- Swetha Perabattula S
- Yavuz S
- Pradhan R

journal: Journal of the Endocrine Society

journalAbbr: J Endocr Soc

year: 2025

pubdate: 2025 Oct-Nov

volume: 9

issue: Suppl 1

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12543815/

Record 240

pmcid: PMC12543589

pmid:

doi: 10.1210/jendso/bvaf149.1463

title: MON-136 BIUXX.

authors

- Sharma R

journal: Journal of the Endocrine Society

journalAbbr: J Endocr Soc

year: 2025

pubdate: 2025 Oct-Nov

volume: 9

issue: Suppl 1

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12543589/

Record 241

pmcid: PMC12094839

pmid: 40406639

doi: 10.1055/s-0045-1806807

title: Multidisciplinary Team Care: Adolescent Breast.

authors

- Mohan VC
- Wang DS

- Skochdopole AJ
- Dvoracek LA

journal: Seminars in plastic surgery

journalAbbr: Semin Plast Surg

year: 2025

pubdate: 2025 May

volume: 39

issue: 2

pages: 89-96

abstract: Poland syndrome, tuberous breast deformity, gynecomastia, and macromastia are among the most common breast disorders affecting adolescents. While the etiology and physical manifestations may differ between each condition, they all may impose psychosocial stress necessitating evaluation and treatment. A multidisciplinary team is essential to providing comprehensive care specific to an adolescent population. The purpose of this chapter is to discuss common adolescent breast deformities necessitating plastic surgeon involvement, and the components of a successful multidisciplinary adolescent breast care team.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12094839/

Record 242

pmcid: PMC12052082

pmid: 40126888

doi: 10.1289/EHP14478

title: Mycoestrogen Exposure during Pregnancy: Impact of the ABCG2 Q141K Variant on Birth and Placental Outcomes.

authors

- Rivera-Núñez Z
- Kinkade C
- Brinker A
- Zhang R
- Buckley B
- Brunner J
- Ohman-Strickland P
- Qiu X
- Qasem RJ
- Fallon JK
- Smith PC
- Miller RK
- Salafia CS
- O'Connor TG
- Aleksunes LM
- Barrett ES

journal: Environmental health perspectives

journalAbbr: Environ Health Perspect

year: 2025

pubdate: 2025 May

volume: 133

issue: 5

pages: 57001

abstract: BACKGROUND: Zearalenone (ZEN) is an estrogenic mycotoxin ("mycoestrogen") that contaminates global grain crops leading to detectable concentrations of ZEN and its metabolites, including the synthetic version α -zearalanol (also called zeranol; ZER), in human populations. Despite in vitro and in vivo animal evidence of endocrine disruption by ZEN, there has been limited investigation in humans. OBJECTIVES: To examine markers of fetal growth following prenatal exposure to ZEN and evaluate the role of the placental efflux transporter BCRP/ ABCG2 in protecting against ZEN's potential fetoplacental toxicity. METHODS: Placentas were collected from participants (n = 271) in the Understanding Pregnancy Signals and Development cohort (Rochester, New York, USA). Placental ZEN and its metabolites were analyzed from tissue samples using HPLC-MS. Birth weights and placental weights were obtained from medical records and direct measurement, respectively; fetoplacental weight ratio (FPR) was calculated by dividing birth weight by placental weight. Covariate-adjusted generalized linear regression models were used to examine ZEN, ZER, and total mycoestrogens (sum of ZEN, ZER, and their metabolites) in relation to birth length, birth weight, placental weight and FPR. We additionally stratified models by infant sex and ABCG2 C421A (Q141K) genotype. RESULTS: Mycoestrogens were detected in 84% of placentas (median ZEN: 0.010 ng / g) and total mycoestrogens were associated with lower FPR [- 0.20 ; 95% confidence interval (CI): - 0.32 , - 0.08], particularly in female infants (- 0.31 ; 95% CI: - 0.52 , - 0.09). Associations with birth weight were inverse and overall nonsignificant. Among the 17% of participants with the reduced function 421A ABCG2 variant (AA or AC), total mycoestrogens were associated with lower birth weight (- 113.5 g ; 95% CI: - 226.5 , - 0.50), whereas in wild-type individuals, total mycoestrogens were associated with higher placental weight (9.9; 95% CI: 0.57, 19.2) and reduced FPR (- 0.19 ; 95% CI: - 0.33 , - 0.05). DISCUSSION: Results from this epidemiological study of prenatal mycoestrogen exposure and perinatal health suggest that mycoestrogens may reduce placental efficiency, resulting in lower birth weight, particularly in female and ABCG2 421A infants. https://doi.org/10.1289/EHP14478.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12052082/

Record 243

pmcid: PMC12525999

pmid: 41097282

doi: 10.3390/molecules30193860

title: Mycotoxin Contamination: Occurrence, Biotransformation, Pathogenic Mechanisms, and Strategies for Nutritional Intervention.

authors

- Yao C
- Ye M
- Wang C
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- Zhang X
- Chai X
- Yu H
- Zhang C
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journal: Molecules (Basel, Switzerland)

journalAbbr: Molecules

year: 2025

pubdate: 2025 Sep 23

volume: 30

issue: 19

pages:

abstract: Mycotoxins, toxic fungal secondary metabolites, exhibit a diverse array of toxicological effects, including hepatotoxicity, carcinogenicity, estrogenicity, immunotoxicity, and neurotoxicity. These toxins cause severe contamination in food, feed, and traditional Chinese medicines (TCMs), threatening global food security and imposing substantial economic burdens. Among over 400 distinct mycotoxins identified to date, aflatoxin B1 (AFB1), ochratoxin A (OTA), and zearalenone (ZEN) stand out for their pervasive contamination and grave toxicities. Upon absorption, these toxins undergo biotransformation into reactive metabolites that exert multifaceted toxicities via mechanisms such as carcinogenesis, estrogenic effects, oxidative stress, inflammation, and abnormal apoptosis, collectively threatening human and livestock health. The application of natural and engineered enterosorbents suppresses intestinal absorption and subsequent bioactivation of mycotoxins, while dietary small-molecule bioactive compounds neutralize post-absorption toxicity via biotransformation intervention and cytoprotective reinforcement, collectively preventing the onset and progression of related diseases. This paper reviews the biosynthetic routes of three representative mycotoxins (AFB1, OTA, and ZEN), along with their biotransformation and underlying pathogenic mechanisms. Furthermore, nutritional intervention approaches targeting the underlying mechanisms to ameliorate mycotoxin-induced damage are discussed. This review not only provides valuable insights for future research on mycotoxin toxicity, but also establishes a theoretical foundation for utilizing dietary strategies to counteract mycotoxin-induced physical damage.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12525999/

Record 244

pmcid: PMC12549669

pmid: 41141691

doi: 10.3389/fsurg.2025.1667856

title: NLR and BMI are independent predictors of postoperative drainage volume in macromastia patients following reduction mammoplasty.

authors

- Wu C
- Zhao L
- Wang J
- Liang H
- Tao Y

journal: Frontiers in surgery

journalAbbr: Front Surg

year: 2025

pubdate: 2025

volume: 12

issue:

pages: 1667856

abstract: BACKGROUND: Accurate prediction of post-reduction mammoplasty drainage volume is critical for optimizing postoperative care and reducing complication risks in patients with macromastia. This study aimed to identify key predictors of total postoperative drainage volume. We further investigated whether these predictors

demonstrate consistent effects across diverse populations or subgroups, thereby providing evidence to support personalized management of postoperative drainage. METHODS: Clinical data from 69 macromastia patients were analyzed, including preoperative and postoperative variables such as body mass index (BMI), neutrophil-to-lymphocyte ratio (NLR), lymphocyte-to-monocyte ratio (LMR), and postoperative differential blood cell counts (e.g., postoperative neutrophils, lymphocytes, and monocytes). Data were summarized using descriptive statistics. Variables significantly associated with total drainage volume were screened via Spearman's correlation analysis. Univariate and multivariate regression analyses were subsequently performed to identify independent predictors. Additionally, stratified subgroup analyses based on BMI and age were conducted to assess the consistency of predictor effects. RESULTS: Univariate and correlation analyses revealed significant positive associations between total drainage volume and both BMI (Spearman's ρ = 0.564, P < 0.0001) and postoperative NLR (Spearman's ρ = 0.506, P < 0.0001). Multivariate regression confirmed BMI (P < 0.001) and postoperative NLR (P = 0.033) as independent and significant predictors of postoperative drainage volume. Furthermore, stratified analyses demonstrated consistent predictive effects for BMI and postoperative NLR across all BMI and age subgroups (P < 0.05), with no significant heterogeneity observed. CONCLUSION: This study identifies BMI and postoperative NLR as independent predictors of total postoperative drainage volume, highlighting their clinical utility. The consistent predictive performance of these factors across BMI and age subgroups supports their broad applicability. These findings provide evidence-based support for personalized drainage management strategies and offer critical insights for clinical practice.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12549669/

Record 245

pmcid: PMC12854343

pmid: 41625644

doi: 10.4103/jehp.jehp_996_25

title: Oral health status during pregnancy and evaluation of a maternal oral screening tool: A multicenter cross-sectional study.

authors

- Sultana R
- Tuli A
- Awinashe M
- Sayed AJ
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journal: Journal of education and health promotion

journalAbbr: J Educ Health Promot

year: 2025

pubdate: 2025

volume: 14

issue:

pages: 564

abstract: BACKGROUND: Pregnancy is associated with several transient hormonal changes that not only affect the patient's health but also their relations with others in their surroundings. The alterations in the hormonal levels may affect the oral as well as systemic health of the individual during pregnancy. The present study aimed to evaluate oral health during pregnancy using a maternal oral screening (MOS) tool. MATERIALS AND METHODS: This multicenter, cross-sectional, prospective study was conducted in the Gynaecology outpatient

department after obtaining Ethical clearance from the Institutional Ethics Committee. The total sample size was 100 pregnant women who were selected in a random manner. The oral health of pregnant female subjects was assessed using a "14-item" OHIP questionnaire along with clinical oral examination. The study data were analyzed using the statistical software SPSS version 20.0 (IBM, NY, US). Descriptive statistics data were observed as mean, standard deviation (SD), and percentages. Statistical association evaluation was done by the "Chi-square" statistical tool. ' P ' values lower than 0.05 were statistically significant. RESULTS: On analyzing the MOS tool, 23.1% had dental problems, whereas 54% visited a dentist during the past year. 34% subjects were "at risk" of poor oral health. The analytical tools reported high sensitivity (75-90%) and low specificity (13-18%) of the MOS tool for the detection of pregnant women who are at risk of developing poor oral health. On comparing the MOS tool with OHIP-14, Item 1 of MOS demonstrated the highest sensitivity (84.3%), whereas Item 2 demonstrated only 52.2% specificity. On combining item numbers 1 and 2, sensitivity was found to increase up to 95.7% but a decrease in specificity was noted. On comparing the MOS tool and clinical oral/dental examination, combining items 1 and 2 resulted in the highest sensitivity (97.2%). On combining items 1 and 2 of the MOS tool, low specificity (12%) for dental assessment and OHIP-14 (17.1%) was obtained. CONCLUSION: The MOS tool can be used for evaluating the oral health status in pregnant females. The present study highlights the importance of conserving oral health during pregnancy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12854343/

Record 246

pmcid: PMC11813068

pmid: 39928792

doi: 10.1097/MD.0000000000040585

title: Ovarian cancer causing hyperprolactinemia: A case report and narrative review.

authors

- Šakinienė S
- Veličkienė D

journal: Medicine

journalAbbr: Medicine (Baltimore)

year: 2025

pubdate: 2025 Feb 7

volume: 104

issue: 6

pages: e40585

abstract: The most common cause of hyperprolactinemia is prolactinoma. In addition, it is necessary to exclude potential physiological and pharmacological factors as well as health disorders to determine the cause of hyperprolactinemia. However, few studies have linked elevated prolactin (PRL) levels to ovarian cancer (OC). OC cells can ectopically release PRL, which then attaches to PRL receptors (PRLRs) in ovarian tissue and initiates signaling cascades that induce OC carcinogenesis. Therefore, we can consider PRL as a biomarker or tumorigenesis factor for OC. Furthermore, both PRL and PRLRs are potential therapeutic targets. A 50-year-old female presented with complaints of breast enlargement, soreness, and hyperprolactinemia, in addition to advanced OC. Hyperprolactinemia along with advanced high-grade serous ovarian carcinoma. Due to the patient's fear of confined spaces, magnetic resonance imaging of the pituitary gland under general anesthesia was prescribed to rule out pituitary pathology. Magnetic resonance imaging was not performed due to the deterioration of the underlying condition, and the patient died 2.5 years after the diagnosis of OC. Hyperprolactinemia caused by OC is a rare condition for which there is a lack of literature and case studies. PRL produced by OC tissue binds to PRLRs in an autocrine or paracrine manner, initiating signaling cascades that induce OC tumorigenesis. In combination with other biomarkers, PRL may serve as a biomarker for OC. To establish the relation between OC and elevated PRL levels, additional large-scale population studies are required, with diagnostic and treatment procedures coming first.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11813068/

Record 247

pmcid: PMC11937744

pmid: 39835610

doi: 10.1111/ans.19407

title: Paediatric breast disease and developmental breast cysts - reflection on 20 years of experience.

authors

- Grant K
- Bochner M

journal: ANZ journal of surgery

journalAbbr: ANZ J Surg

year: 2025

pubdate: 2025 Mar

volume: 95

issue: 3

pages: 497-502

abstract: INTRODUCTION: Presentation with breast symptoms in the paediatric population is common but there is little existing literature on the workup and management of breast disease in this population. METHODS: Retrospective series of 140 cases of breast disease in the paediatric population managed by a single surgeon in Adelaide, South Australia between 2004 and 2024. Review of patient demographics, presentation, investigation, management and outcomes of various breast symptoms and pathologies using descriptive analysis. RESULTS: We reviewed 140 cases including 135 females and 5 males, aged 8-18 years (median = 15). The most common reason for presentation was breast lump (n = 86, 61%), followed by large breasts (n = 13, 9%), asymmetry (n = 11, 8%), abnormal development (n = 8, 6%). Most patients were investigated with ultrasound (n = 101, 71%) and FNA biopsy was performed in cases of non-diagnostic ultrasound or to distinguish fibroadenoma from phyllodes tumour (n = 17, 12%). There was a single case of malignancy and the most common diagnoses were developmental breast cyst (n = 36, 26%) and fibroadenoma (n = 33, 24%). Gynaecomastia was the only diagnosis in males (n = 5, 4%). Majority of patients were managed non-operatively (n = 101, 72%), indications for surgery included fibroadenoma (n = 18, 13%), breast hypertrophy (n = 8, 6%) or abnormal breast development (n = 4, 3%). CONCLUSION: We reviewed 140 cases of breast disease in the paediatric population over a 20-year period. Malignancy in this population is rare but requires exclusion with history, examination and ultrasound where appropriate. Developmental breast cysts are the most common pathology, require early identification to avoid unnecessary invasive investigation and should be managed expectantly where possible.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11937744/

Record 248

pmcid: PMC12648882

pmid: 41291554

doi: 10.1186/s12887-025-06314-z

title: Parent-adolescent communication on reproductive and sexual issues and its determinants among parents in Gondar town, Northwest Ethiopia: a mixed-methods study.

authors

- Gedef GM
- Andualem F

- Takelle GM
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journal: BMC pediatrics

journalAbbr: BMC Pediatr

year: 2025

pubdate: 2025 Nov 25

volume: 25

issue: 1

pages: 958

abstract: BACKGROUND: Sexual and reproductive health problems among adolescents in developing countries are increasing significantly. Assessing parent-adolescent communication is crucial for identifying barriers and fostering supportive relationships to help adolescents make informed health decisions. Therefore, this study aimed to evaluate parent-adolescent communication on reproductive and sexual health issues and its determinants. METHODS: A mixed-methods approach combining both quantitative and qualitative was conducted in Gondar town from January 1 to February 28, 2022. The quantitative data were collected from 673 parents of adolescents aged 10-19 years using an interviewer-administered questionnaire, while qualitative data were gathered through seventeen in-depth interviews. Data were analyzed using SPSS version 25 and qualitative data manually analyzed using thematic analysis. Finally, the qualitative and quantitative findings were triangulated for comprehensive insights. RESULTS: Only 266 (39.5%) respondents reported having discussions on reproductive and sexual health issues with their adolescents. A father's occupation as a merchant (AOR = 0.45; 95% CI: 0.29, 0.69), a family size of more than five (AOR = 0.39; 95% CI: 0.23, 0.66), good knowledge (AOR = 4.45; 95% CI: 2.90, 6.84), and a positive attitude toward sexual and reproductive health (AOR = 1.62; 95% CI: 1.15, 2.28) were predictors. Furthermore, qualitative findings revealed that parent-adolescent communication occurs infrequently and is influenced by sociocultural norms, and parents' awareness of sexual and reproductive health issues. CONCLUSIONS: Nearly two in five parents engaged in parent-adolescent discussions on reproductive health issues. These discussions were influenced by a lack of parental knowledge, sociocultural norms, and concerns that such conversations might encourage premarital sex. To address these challenges, national reproductive health programs should focus on community-centered campaigns to raise awareness and equip parents with comprehensive knowledge, while school-based programs should promote healthy adolescent behaviors through age-appropriate health education and life skills training.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12648882/

Record 249

pmcid: PMC12521779

pmid: 41103835

doi: 10.7759/cureus.92316

title: Peripheral Precocious Puberty Due to Autonomous Gonadal Activation: A Multicenter Experience.

authors

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journal: Cureus

journalAbbr: Cureus

year: 2025

pubdate: 2025 Sep

volume: 17

issue: 9

pages: e92316

abstract: INTRODUCTION: Peripheral precocious puberty (PP) is far less commonly encountered compared to central precocious puberty (CPP) in pediatric endocrine practice. Long-standing non-diagnosis may cause rapidly advancing bone age, leading to CPP and resultant short stature. We aimed to report the clinical profile and current Indian experience in the management of children with peripheral PP due to autonomous gonadal activation. METHODS: This multicentric retrospective study reports data on 23 children (20 girls) presenting with peripheral PP as a result of autonomous gonadal activation from eight pediatric endocrine centers across India. Their clinicodemographic, anthropometric, and laboratory measurements were reviewed. RESULTS: The mean ± SD chronological age at the time of presentation was 4.9 ± 2.0 years, and the mean bone age was 7.6 ± 2.6 years. Nine (39%) children were tall for mid-parental height. Thirteen (57%) children were diagnosed with McCune Albright syndrome (MAS), one boy (4%) with familial male-limited PP, and nine (39%) with functional ovarian cysts. Ninety-five percent of girls had menarche as their presenting complaint, with mean age at menarche being 4.6 ± 2.3 years. Ovarian cysts were present in 16 girls, of whom seven (43.8%) had MAS. PP in seven (30%) children progressed to CPP, with mean age of CPP being 6.6 ± 2.1 years. Letrozole was the primary drug of choice, while leuprolide acetate was added in children who progressed to CPP. CONCLUSION: We report the clinical profile of children with peripheral PP due to autonomous gonadal activation. Clinical diagnosis and timely intervention to halt the progression of puberty and prevent early epiphyseal fusion, thereby improving final height, along with close follow-up, are vital in the management of these disorders.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12521779/

Record 250

pmcid: PMC12141334

pmid: 40487002

doi: 10.3389/fsoc.2025.1544169

title: Poverty and large family size are pivotal risk factors for child marriage in Ethiopia: evidence from a matched case-control study.

authors

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journal: Frontiers in sociology

journalAbbr: Front Sociol

year: 2025

pubdate: 2025

volume: 10

issue:

pages: 1544169

abstract: BACKGROUND: Child marriage is a marriage carried out before the age of 18 years old which is influenced by numerous cultural, social, and economic factors, and it is a source of gender inequality, violence against women, and various maternal and child health problems. Therefore, this study aimed to identify risk factors of child marriage in the West Guji zone of southern Oromia, Ethiopia. METHODS: A community-based 1:2 matched case-control study design was conducted from April 1 to May 5, 2024. The purposive sampling technique was used to select 120 pairs (120 cases and 240 controls). Age was selected as a matching variable, and for each case, two best-matched controls were selected. The questionnaires were administered through a face-to-face interview. Both binary and multivariable conditional logistic regressions were conducted to determine independent determinants of child marriage. In the multivariable model, statistical significance was established at $p < 0.05$. The strength of the association was reported by the matched adjusted odds ratio (mAOR) with 95% CI. RESULTS: Women from medium family size (4-6 members) were nearly four times more likely to be married early as compared to women from small family size (≤ 3 family members) (mAOR: 3.87, 95% CI = 1.56-9.55). The odds of early marriage were five-fold greater among women from larger families (mAOR: 5.09, 95% CI = 1.53-16.90). The odds of being married younger than 18 years were nearly three times greater among women who were not aware of the legal age of marriage (mAOR: 2.92, 95% CI = 1.27-6.68). Women whose marriage decision was made by others were two times more likely to be married early (mAOR: 2.47, 95 CI = 1.30-4.71). The risk of marriage under 18 years of age was seven-fold greater among women from the poor parental wealth category than among those from the rich category (mAOR: 7.65, 95%CI = 2.48-13.07). CONCLUSION: Poor family wealth statuses, larger parental family size, not knowing the legal marital age, and marriage decision by others, were risk factors for child marriage. As a result, to stop child marriage in rural Ethiopia, policies and strategies that take into account these factors should be developed and put into practice.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12141334/

Record 251

pmcid: PMC12049125

pmid: 40322033

doi: 10.2147/DDDT.S517485

title: Preclinical Investigations of a Novel PEGylated Long-Acting Human Growth Hormone.

authors

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journal: Drug design, development and therapy

journalAbbr: Drug Des Devel Ther

year: 2025

pubdate: 2025

volume: 19

issue:

pages: 3497-3507

abstract: PURPOSE: The daily administration of recombinant human growth hormone (rhGH) replacement therapies via subcutaneous (SC) injections can be burdensome for patients. Long-acting Growth hormone (GH)

formulation could reduce injection frequency, and it will be expected to increase treatment adherence. ZHB111 is a PEGylated rhGH that requires sc injection every two weeks. This study aims to examine the pharmacokinetics (PK), pharmacodynamics (PD), safety, and tolerability of multiple sc doses of ZHB111. DESIGN AND METHODS: In the PK study, naïve cynomolgus monkeys were divided into four groups with 3/sex/group. Each group received single subcutaneous administration at 0 (control article, Sodium chloride injection), 0.3, 1.0 and 3.0 mg/kg, respectively. To access safety and PD profile, cynomolgus monkeys were randomly assigned to 4 groups of 5/sex/group, and were dosed with ZHB111 once weekly for 4 times by subcutaneous injection at 0 (control article, Sodium chloride injection), 1, 5 or 20.6 mg/kg/dose. Tissue distribution was accessed by administering a single subcutaneous dose of ZHB111 to Sprague-Dawley Rats. RESULTS: ZHB111 exhibits a significant difference in serum half-life (in monkeys) across varying dosages, with a trend of decreasing half-life as the dosage increases, ranging from 110-30 h. No significant differences were observed between different genders at the same dosage level. After subcutaneous administration of 0.3mg/kg of ZHB111, the mean values of IGF-1 in male animals ranged among 594.93~1294.98ng/mL at various time points, while those in female animals were within the range of 394.15~770.01ng/mL. CONCLUSION: The tolerability of ZHB111 was demonstrated in both rat and monkey models, with only minimal adverse effects observed. These findings support the continued clinical development of ZHB111 as a novel treatment for growth hormone deficiency (GHD) in both children and adult patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12049125/

Record 252

pmcid: PMC12051815

pmid: 40330183

doi: 10.18502/ijph.v54i3.18258

title: Predisposing Factors for Idiopathic Granulomatous Mastitis: A Large Prospective Multicentric Study.

authors

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journal: Iranian journal of public health

journalAbbr: Iran J Public Health

year: 2025

pubdate: 2025 Mar

volume: 54

issue: 3

pages: 645-653

abstract: BACKGROUND: Idiopathic granulomatous mastitis (IGM) is a chronic breast disease with unknown pathophysiological and clinical aspects. Therefore, we designed this multi-center prospective case-control study to explore and clarify the risk factors with an acceptable sample size. METHODS: From March 2021 to December 2023, five hundred-four women with a histologically proven IGM were entered into the IGM group and 504 women with healthy breasts were included in the control group across nine centers. Participants' information was obtained by trained staff. Multivariate binary logistic regression was used to estimate the odds ratio (OR) and 95% confidence intervals (CI) for the associations between variables and IGM. RESULTS: The mean age of all participants was 34.92 ± 7.12 yr. The previous history of diseases as a whole, was significantly higher in the IGM group (33.1%) compared with the control group (26.6%). Logistic regression showed that previous lactation (OR= 7.51, 95% CI= 2.37-23.77) and a positive history of diabetes (OR = 4.23, 95% CI= 1.32-13.51) had a positive association with IGM, while OCP use (OR = 0.70, 95% CI= 0.52-0.93) was associated with IGM reversely. CONCLUSION: The history of breastfeeding and not its duration is associated with a 7-fold increase in the rate of IGM. Furthermore, previous history of diabetes is a risk factor, while OCP use is a protective factor against IGM. We are designing another study to further investigate the relationship between breastfeeding, milk stasis, and IGM, based on our findings.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12051815/

Record 253

pmcid: PMC12542117

pmid: 41121440

doi: 10.1186/s40644-025-00941-6

title: Pregnancy associated breast cancer: correlation between parity and ultrasonic characteristics of tumors & background echotexture.

authors

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journal: Cancer imaging : the official publication of the International Cancer Imaging Society

journalAbbr: Cancer Imaging

year: 2025

pubdate: 2025 Oct 21

volume: 25

issue: 1

pages: 119

abstract: OBJECTIVE: To investigate the characteristics of pregnancy associated breast cancer (PABC) and correlation of parity with ultrasonic tumor and background echotexture, based on the analysis of diagnostic efficiency. METHODS: The ultrasonic images of 184 female patients with PABC and 47 female patients with

benign lesions, all of whom were pregnant or within one year postpartum at diagnosis and underwent surgery for their breast conditions in our center from January 2016 to December 2023, were retrospectively analyzed. Ultrasound diagnostic performance was assessed by the receiver operating characteristic (ROC) curve and the area under the ROC curve (AUROC). Breast ultrasound background echotexture was classified according to two criteria: the first classification was homogeneous-fat, homogeneous-fibroglandular, and heterogeneous, the other classification was hypoechoic dominated and hyperechoic dominated. The correlation between parity and ultrasonic characteristics (tumor and background echotexture) was analyzed. RESULTS: The AUROC of ultrasound diagnosis for PABC was 0.939 (95%CI: 0.910-0.969). PABC tumors were predominantly irregular in shape, uncircumscribed, hypoechoic, heterogeneous, and solid. Compared with primiparous women, the PABC tumors were larger in multiparous women (4.00± 3.30cm vs. 3.09 ± 2.01cm, P = 0.037), with a higher rate of lymphatic metastasis (57.27% vs. 41.89%, P = 0.041). In terms of background echotexture, heterogeneous echotexture and hypoechoic dominated were more frequently observed in multiparas than in primiparas (52.73% vs. 37.84%, P= 0.047; 49.09% vs. 32.43%, P = 0.025). Multivariate analysis further indicated multiparous women were more likely to have heterogeneous (OR = 2.241, 95% CI:1.032-4.867, P= 0.041) and hypoechoic dominated (OR = 2.064, 95% CI:1.045-4.077, P = 0.037) breast ultrasound backgrounds than primiparous women, adjusted for confounders. CONCLUSIONS: Ultrasound is effective for diagnosing PABC. Multiparas may show a more heterogeneous and predominantly hypoechoic background echotexture, an increased risk of lymphatic metastasis, and larger tumors.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12542117/

Record 254

pmcid: PMC11879532

pmid: 40046611

doi: 10.1155/proc/8165686

title: Preliminary Evidence on Safety and Clinical Efficacy of Luteolin for Patients With Prostate Cancer Under Active Surveillance.

authors

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journal: Prostate cancer

journalAbbr: Prostate Cancer

year: 2025

pubdate: 2025

volume: 2025

issue:

pages: 8165686

abstract: Background: A need exists for effective treatments for prostate cancer (PCA) due its re-emergence following androgen deprivation therapy, a major clinical problem. In a previous study, we presented evidence on the chemopreventive and chemotherapeutic potential of luteolin, a flavonoid, in PCA including castration-resistant PCA. In this single-arm phase I study, we clinically examined the safety of the oral intake of luteolin in patients under active surveillance (AS). Methods: Between March and September in 2022, five patients with low-intermediate risk PCA and under AS were treated daily with 50 mg of oral luteolin for six months. We investigated the efficacy of oral luteolin in oncological outcomes and any adverse events (AEs) and examined prostate and blood specimens. Results: The median age of patients was 68 years (range: 60-78), and the median initial prostate-specific antigen level was 9.5 ng/mL. All patients were under AS without rapid progression. After treatment with luteolin, AEs were not noted in any patients for six months. All patients underwent a protocol biopsy. Of these, two patients showed a favorable response, one patient had stable disease, and two patients showed disease progression; robot-assisted radical surgery was subsequently performed for the latter. Immunohistochemical analysis revealed decreased expression of androgen receptor and NKX3.1 in noncancerous lesions after luteolin treatment. In addition, quantitative reverse transcription-PCR revealed that serum micro(mi)RNA expression in serum and prostate gland, including miR-29 and miR-30, tended to be upregulated after luteolin treatment compared with during the pretreatment phase. Conclusions: Our small phase I study of men with PCA suggests that daily treatment with 50 mg of an oral supplement of luteolin is safe and effective with regard to oncological outcomes, particularly in patients under AS.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11879532/

Record 255

pmcid: PMC12609238

pmid: 41226997

doi: 10.3390/jcm14217601

title: Preoperative MRI-Based 3D Segmentation and Quantitative Modeling of Glandular and Adipose Tissues in Male Gynecomastia: A Retrospective Study.

authors

- Shi Z
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journal: Journal of clinical medicine

journalAbbr: J Clin Med

year: 2025

pubdate: 2025 Oct 27

volume: 14

issue: 21

pages:

abstract: Background: This study aimed to explore the application value of magnetic resonance imaging (MRI)-based three-dimensional segmentation and reconstruction technology for spatial structural identification and volumetric quantification of glandular and adipose tissues in bilateral gynecomastia (GM) patients undergoing surgical treatment, hoping to provide precise imaging data to support clinical surgical decision-making. Methods: A retrospective analysis was performed on preoperative MRI images and general clinical data of 52 patients with bilateral gynecomastia at the patient level (bilateral totals, N = 52) who underwent surgical treatment in the

Department of Aesthetic and Reconstructive Breast Surgery, Plastic Surgery Hospital of Chinese Academy of Medical Sciences, from March 2023 to September 2024. All images were acquired using a SIEMENS Aera 1.5 T MRI scanner with T1-weighted three-dimensional fat-suppressed sequence (t1_fl3d_tra_spair). Semi-automatic segmentation and active contour modeling (Snake model) using ITK-SNAP 4.0 software were employed to independently identify glandular and adipose tissues, reconstruct accurate three-dimensional anatomical models, and quantitatively analyze tissue volumes. Results: The MRI-based three-dimensional segmentation and reconstruction method accurately distinguished glandular and adipose tissues in male breasts, establishing precise three-dimensional anatomical models with excellent reproducibility and operational consistency. Among the 52 patients with bilateral gynecomastia, glandular tissue volume exhibited a markedly non-normal distribution, with a median of 6.11 cm 3 (IQR, 3.03-12.98 cm 3). Adipose tissue volume followed a normal distribution with a mean of 1348.84 ± 494.97 cm 3 . The total breast tissue volume also showed a normal distribution, with a mean of 1361.97 ± 496.83 cm 3 . The proportion of glandular tissue in total breast volume was non-normally distributed with a median of 0.50% (IQR, 0.27-1.21%), while the proportion of adipose tissue was also non-normally distributed with a median of 99.50% (IQR, 98.79-99.73%). Conclusions : MRI combined with computer-assisted three-dimensional segmentation and reconstruction technology efficiently and accurately achieves spatial identification, three-dimensional modeling, and volumetric quantification of glandular and adipose tissues in patients with bilateral gynecomastia. It objectively reveals the spatial compositional characteristics of male breast tissues. This approach provides precise, quantitative data for clinical decision-making regarding surgical treatment of gynecomastia, featuring robust standardization and strong clinical applicability.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12609238/

Record 256

pmcid: PMC12313775

pmid: 40244528

doi: 10.1007/s40618-025-02583-8

title: Prevention of bicalutamide-induced breast events in patients with prostate cancer: a meta-analysis of randomized controlled trials.

authors

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journal: Journal of endocrinological investigation

journalAbbr: J Endocrinol Invest

year: 2025

pubdate: 2025 Jul

volume: 48

issue: 7

pages: 1659-1668

abstract: PURPOSE: This study aimed to quantitatively assess the effectiveness of tamoxifen, anastrozole, and radiotherapy in preventing bicalutamide-induced breast events-specifically gynecomastia and breast pain-in

patients with prostate cancer. METHODS: A systematic review and meta-analysis of randomized controlled trials (RCTs) was conducted according to PRISMA-P guidelines. A comprehensive search was performed in PubMed, Scopus, and Web of Science for English-language studies without temporal restrictions. Studies were included if they involved prostate cancer patients treated with bicalutamide receiving preventive interventions (tamoxifen, anastrozole, or radiotherapy) compared to bicalutamide alone (or bicalutamide plus placebo/sham). Data extraction focused on the incidence of gynecomastia and breast pain, and study quality was assessed using the Jadad scale. Risk ratios (RR) with 95% confidence intervals (CI) were calculated using fixed- or random-effects models, and heterogeneity was evaluated with the I^2 statistic. Publication bias was explored via funnel plots and the trim-and-fill method. RESULTS: Nine RCTs met the inclusion criteria. Tamoxifen significantly reduced the risk of breast events by 82% (RR: 0.18, 95% CI: 0.08-0.38 for gynecomastia and RR: 0.18, 95% CI: 0.07-0.43 for breast pain). Radiotherapy reduced gynecomastia risk by 52% (RR: 0.48, 95% CI: 0.38-0.59) and breast pain by 34% (RR: 0.66, 95% CI: 0.48-0.90). Anastrozole did not show significant benefit. CONCLUSION: Tamoxifen appears to be the most effective strategy for preventing bicalutamide-induced breast events, with radiotherapy serving as a viable alternative, and anastrozole offering no benefit. Further large-scale, high-quality studies are needed to confirm these findings and refine preventive treatment recommendations.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12313775/

Record 257

pmcid: PMC11857216

pmid: 40881979

doi: 10.1159/000543952

title: Primary Breast Lymphoma in a Young Female: Non-Hodgkin's Diffuse Large B-Cell Lymphoma in the Presence of Intact Breast Prostheses.

authors

- Al-Bitar A
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journal: Case reports in oncology

journalAbbr: Case Rep Oncol

year: 2025

pubdate: 2025 Jan-Dec

volume: 18

issue: 1

pages: 271-277

abstract: INTRODUCTION: Primary breast lymphoma (PBL) is a rare non-Hodgkin lymphoma, predominantly presenting as diffuse large B-cell lymphoma (DLBCL) and primarily affecting the breast, often misdiagnosed due to its nonspecific symptoms resembling breast carcinoma. Breast implants are among the most frequently used medical devices for esthetic and reconstructive purposes. CASE PRESENTATION: We present a 24-year-old female with a growing breast mass who was diagnosed with PBL (non-Hodgkin's DLBCL). She underwent successful R-CHOP chemotherapy, resulting in complete remission. Follow-up imaging showed no distant metastasis or significant residual disease, with silicone implants intact and no pathological uptake. CONCLUSION: PBL is a rare malignancy that primarily affects postmenopausal women, with diagnosis relying on radiologic investigations and histopathology due to its clinical and imaging similarities to breast carcinoma. Treatment is controversial and often involves rituximab-based chemotherapy, intrathecal methotrexate, and radiotherapy, with improved outcomes observed in recent years.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11857216/

Record 258

pmcid: PMC12687399

pmid: 41121676

doi: 10.47717/turkjsurg.2025.2025-6-14

title: Proactive fat grafting from the breast area in gynecomastia surgery: Impact on the prevention of contour irregularities and patient satisfaction.

authors

- Ersan M
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journal: Turkish journal of surgery

journalAbbr: Turk J Surg

year: 2025

pubdate: 2025 Dec 8

volume: 41

issue: 4

pages: 374-382

abstract: OBJECTIVE: Contour irregularities are among the most significant complications that may occur after gynecomastia surgery and they can adversely affect aesthetic outcomes. This study evaluated the use of proactively harvested autologous fat grafts from the same surgical field prior to gynecomastia surgery for the treatment of intraoperative contour deformities, as well as the impact of this approach on patient satisfaction. MATERIAL AND METHODS: A retrospective evaluation was conducted on 24 male patients who were diagnosed with gynecomastia between April 2023 and March 2025 through physical examination, endocrinology consultation, and breast ultrasonography, who subsequently underwent surgical treatment. Prior to surgery, autologous fat grafts were harvested from the breast area in all patients. Intraoperatively identified contour irregularities were treated with same-session fat injection using the previously harvested grafts. All patients were followed up with ultrasonography and digital photography. Additionally, patient satisfaction was assessed using BODY-Q chest module and the chest satisfaction questionnaire. RESULTS: Of the 24 patients, 83% (n=20) presented with pseudogynecomastia and were treated with liposuction alone, while 17% (n=4) had gynecomastia and underwent liposuction combined with gland excision. Intraoperative contour irregularities were detected in 9 patients (37.5%), in whom an average of 8 cc (interquartile range 7-10) of fat was injected. Over a mean follow-up of 12.4 months, no statistically significant difference in aesthetic satisfaction was observed between patients with and without fat grafting ($p>0.05$). Both groups reported high satisfaction, and intraoperative contour deformities were successfully corrected. CONCLUSION: Proactively harvesting autologous fat from the breast tissue during gynecomastia surgery may be a safe and practical method for immediate correction of intraoperative contour irregularities. This approach avoids additional donor site morbidity and provides a readily available graft source. While satisfaction outcomes were high in all patients, larger prospective studies are needed to confirm the long-term efficacy and broader applicability of this technique.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12687399/

Record 259

pmcid: PMC12460840

pmid: 40993108

doi: 10.1038/s41538-025-00541-5

title: Prunella vulgaris seed oil alleviates cancer related fatigue through hypothalamic inflammation and CRH regulation.

authors

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journal: NPJ science of food

journalAbbr: NPJ Sci Food

year: 2025

pubdate: 2025 Sep 24

volume: 9

issue: 1

pages: 194

abstract: Cancer-related fatigue (CRF) is a common symptom in cancer patients, affecting their quality of life. Prunella vulgaris and its seed oil (PVSO) are edible-medicinal foods with bioactive compounds that may alleviate CRF. The present study aimed to test the effects of PVSO on CRF and underlying mechanisms. We found PVSO improved fatigue-like behaviors in both LPS-induced and tumor-bearing models. In the breast cancer tumor-bearing animals, PVSO reduced tumor growth, and suppressed serum and hypothalamic inflammatory cytokines (TNF-α, IL-1β, and IL-6) and NF-κB signaling. Furthermore, the hypothalamic corticotropin-releasing hormone (CRH) expression and serum CRH/cortisol levels dampened in tumor-bearing animals were normalized by PVSO. Our results suggest that PVSO exhibits potent anti-CRF effects likely via attenuation of cancer-induced hypothalamic inflammation and subsequent normalization of the dysregulated HPA axis. PVSO may be developed as a functional food for breast cancer-related fatigue.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12460840/

Record 260

pmcid: PMC12205930

pmid: 40585415

doi: 10.5599/admet.2685

title: Rapid detection of illegal selective androgen receptor modulators in unregistered supplements using a combination of selected solid-state analytical methods.

authors

- Jendrzejewska I
- Cehlarik L
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journal: ADMET & DMPK

journalAbbr: ADMET DMPK

year: 2025

pubdate: 2025

volume: 13

issue: 3

pages: 2685

abstract: BACKGROUND AND PURPOSE: Pharmaceutical crime is becoming an increasingly serious threat. For customs officers and police officers, minimal or no sample preparation before analysis is essential when detecting prohibited compounds/counterfeit products, ideally using on-site analysis. EXPERIMENTAL APPROACH: Unregistered dietary supplements containing anabolic substances, specifically selective androgen receptor modulators (SARMs) such as andarine, ligandrol, ostarine, and testolone became the subject of investigation. Dietary supplements with these SARMs are often found illegally in various fitness centres and can be purchased online. Furthermore, these illegal supplements may not contain the declared SARMs at all or may contain incorrect amounts. Analytical methods such as Raman spectroscopy, differential scanning calorimetry, thermogravimetry, and X-ray powder diffraction were chosen to identify these banned SARMs in the illegal products. KEY RESULTS: Using a combination of these fast and direct analytical techniques, in particular, Raman spectroscopy, it was found that out of 16 samples, SARMs were confirmed in 9 samples, while no testolone (4 samples), ostarine (2 samples), or andarine (1 sample) was reliably identified in 7 samples. CONCLUSION: Overall, almost half of the analyzed samples of unregistered/illegal sports dietary supplements purchased anonymously online in the Slovak Republic with declared content of at least one SARM did not contain what is declared on the label. Thus the combination of several solid-state analytical techniques without complex sample preparation has proven valuable for rapid identification of APIs in supplements.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12205930/

Record 261

pmcid: PMC12708142

pmid: 41398854

doi: 10.1097/MD.0000000000046548

title: Rapidly growing unilateral pseudoangiomatous stromal hyperplasia in a 10-year-old girl: A case report and review of management strategies.

authors

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- Han X

journal: Medicine

journalAbbr: Medicine (Baltimore)

year: 2025

pubdate: 2025 Dec 12

volume: 104

issue: 50

pages: e46548

abstract: RATIONALE: Pseudoangiomatous stromal hyperplasia (PASH) is a benign breast condition characterized by collagen fiber-segmented, blood vessel-like clefts lined by spindle cells, lacking red blood cells.

First described by Vuitch et al in 1986, PASH typically presents with benign imaging features and is often identified incidentally during investigations for other breast conditions. PATIENT CONCERNS: This case study documents a rare instance of unilateral breast PASH in a 10-year-old girl, which demonstrated rapid growth over a short period, necessitating surgical excision. The patient presented with a rapidly enlarging, painless right breast mass over 2 months. Clinical examination revealed a soft, poorly defined mass measuring approximately 15 × 15 cm with overlying venous engorgement. Imaging studies, including ultrasound and magnetic resonance imaging, showed a low echogenicity mass with irregular borders and non-uniform enhancement. DIAGNOSES: Histological features consistent with PASH. INTERVENTIONS: Surgical intervention confirmed a well-circumscribed mass with intact capsule. OUTCOMES: Postoperative follow-up over 1 year revealed no recurrence. LESSONS: PASH, typically diagnosed incidentally, can exhibit rapid growth and variable imaging characteristics, sometimes mimicking malignant conditions. Its occurrence in children and adolescents is uncommon, with most cases presenting as benign. The association with hormonal factors, observed in adults and those receiving hormonal treatments, suggests a potential link with endocrine changes. In conclusion, while PASH generally has a favorable prognosis, large or rapidly growing lesions in pediatric patients should be managed surgically to prevent potential functional and cosmetic issues.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12708142/

Record 262

pmcid: PMC12060906

pmid: 40352672

doi: 10.1039/d5md00027k

title: Revisiting the role of steroidal therapeutics in the 21st century: an update on FDA approved steroidal drugs (2000-2024).

authors

- Singh R
- Bansal R

journal: RSC medicinal chemistry

journalAbbr: RSC Med Chem

year: 2025

pubdate: 2025 Jul 16

volume: 16

issue: 7

pages: 2902-2918

abstract: Steroids are biologically active polycyclic compounds that have garnered significant scientific attention due to their distinct physiochemical properties and diverse medical applications. Since their invention more than 90 years ago, steroids have remained the most important and necessary class of regulatory molecules in the evolution process of living creatures and have fascinated scientists due to their broad-spectrum biological activities. Over time, scientific innovations and expanded understanding of mechanisms related to diversified biological activities of steroids have made them cheaper, efficient and more specific therapeutic agents which could be effective in the prevention and cure of numerous diseases like cancer, inflammation, asthma, microbial infection, and many more. However, steroidal drugs remain a double-edged sword having significant therapeutic benefits but with incidence of several adverse effects if used for a longer duration and/or with incorrect dose. Nevertheless, novel treatment approaches such as nanoparticles or liposomal drug delivery, real-time monitoring and the use of artificial intelligence in steroidal therapy outweigh their risk factors and provide an effective and safe treatment with minimum adverse effects. Furthermore, the repurposing of steroids in different diseases, e.g. successful use of dexamethasone or hydrocortisone during COVID-19 pandemic has renewed the interest in steroidal therapeutics. The present review provides an update on FDA approved steroidal drugs during the years 2000-2024, the status of their clinical studies, the challenges offered by steroidal therapy and the future perspectives to counterbalance all these challenges. Moreover, this review also delivers useful data

on the repurposing of steroidal drugs against various diseases along with the novel techniques used for improved steroid delivery.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12060906/

Record 263

pmcid: PMC12544335

pmid:

doi: 10.1210/jendso/bvaf149.1970

title: SAT-121 Van WYK- Grumbach Syndrome: A case report.

authors

- Dash K
- Ayyagari U
- Puran N
- Pandey A

journal: Journal of the Endocrine Society

journalAbbr: J Endocr Soc

year: 2025

pubdate: 2025 Oct-Nov

volume: 9

issue: Suppl 1

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12544335/

Record 264

pmcid: PMC12544090

pmid:

doi: 10.1210/jendso/bvaf149.1799

title: SAT-233 Neonatal Giant Mastauxe: A case report.

authors

- Riaz AA
- Haider R
- Naseer F
- Fatima S
- Khan A
- Mal M
- Qadeer A

journal: Journal of the Endocrine Society

journalAbbr: J Endocr Soc

year: 2025

pubdate: 2025 Oct-Nov

volume: 9

issue: Suppl 1

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12544090/

Record 265

pmcid: PMC12545375

pmid:

doi: 10.1210/jendso/bvaf149.583

title: SAT-726 Nonstandard Hormonal Management of Gynecomastia and Bone Health in a BRCA2-Positive Male on Androgen Deprivation Therapy.

authors

- Kang S
- Foster A
- Bier R

journal: Journal of the Endocrine Society

journalAbbr: J Endocr Soc

year: 2025

pubdate: 2025 Oct-Nov

volume: 9

issue: Suppl 1

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12545375/

Record 266

pmcid: PMC12192740

pmid: 40565151

doi: 10.3390/ijms26125669

title: Severe Insulin Resistance Syndromes: Clinical Spectrum and Management.

authors

- Pliszka M
- Szablewski L

journal: International journal of molecular sciences

journalAbbr: Int J Mol Sci

year: 2025

pubdate: 2025 Jun 13

volume: 26

issue: 12

pages:

abstract: Insulin resistance is a condition wherein cells fail to adequately respond to insulin. It is a prevalent medical condition associated with several diseases, such as type 2 diabetes mellitus, metabolic syndrome, hypertension, obesity, and polycystic ovary syndrome. Insulin resistance may be involved in metabolic disturbances, such as hyperglycemia, hyperinsulinemia, dyslipidemia, hyperuricemia, endothelial dysfunction, elevated inflammatory markers, and a prothrombotic state. Severe insulin resistance syndromes are a heterogeneous group of rare disorders. These disorders are characterized by profound insulin resistance, substantial metabolic abnormalities, and different clinical manifestations and complications. They may be hereditary or acquired, caused by defects in insulin action and cellular responsiveness to insulin. Severe insulin resistance syndromes may also be due to aberrations in adipose tissue function and development. The majority of these disorders are associated with an increased risk of severe complications and mortality. This review aims to summarize the current knowledge on the epidemiology, pathophysiology, complications and prognosis of severe insulin resistance syndromes, as well as to categorize these syndromes by disease process, including defects in insulin receptor, intracellular insulin signaling defects, lipodystrophies, etc.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12192740/

Record 267

pmcid: PMC11726815

pmid: 39813438

doi: 10.1590/0034-7167-2024-0094

title: Social Representations of pregnancy among trans men.

authors

- Pereira DMR
- Araújo EC
- Oliveira SC
- Sousa AR
- Espíndola MMM
- West MGLN
- Ramalho MNA
- Cardoso JC

journal: Revista brasileira de enfermagem

journalAbbr: Rev Bras Enferm

year: 2025

pubdate: 2025

volume: 77Suppl 2

issue: Suppl 2

pages: e20240094

abstract: OBJECTIVE: to understand the meanings and experiences of pregnancy among trans men in light of the Theory of Social Representations. METHODS: this is a qualitative, descriptive and exploratory study, carried out with trans men selected for convenience and availability. Data production took place from September to October 2021, via the Google Meet® platform, based on interviews with a semi-structured script. Interview lexicographic textual analysis was performed using the Reinert method and instrumented by IRAMUTEQ version 7.0. RESULTS: social representations of pregnancy involved a wide field of meanings, in which efforts were articulated to accept opportune and solitary pregnancy, fear of the parturition process and influence of physical and emotional changes. FINAL CONSIDERATIONS: the study reinforces the importance of advanced nursing practice in assisting the pregnancy-puerperal cycle, based on the perspective of comprehensive care, equity in access to services and respect for differences.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11726815/

Record 268

pmcid: PMC12543952

pmid:

doi: 10.1210/jendso/bvaf149.2077

title: SUN-144 Initiating Estradiol Hormone Therapy in a Transgender Woman With Multifocal Idiopathic Localization-Related Epilepsy.

authors

- Sia T
- Chang JJ

journal: Journal of the Endocrine Society

journalAbbr: J Endocr Soc

year: 2025

pubdate: 2025 Oct-Nov

volume: 9

issue: Suppl 1

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12543952/

Record 269

pmcid: PMC11749582

pmid: 39839445

doi: 10.1097/GOX.0000000000006446

title: Surgical Management of Gestational Gigantomastia: A Case Report Highlighting Therapeutic Intervention.

authors

- Lemma MA
- Debodina YS
- Gebremedhin MF
- Aman MK
- Lucero-Prisno DEI

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2025

pubdate: 2025 Jan

volume: 13

issue: 1

pages: e6446

abstract: Gigantomastia is an exceedingly rare condition characterized by extraordinary growth of breasts during pregnancy, and its underlying etiology remains elusive. Although surgical intervention is the primary treatment

modality, there have been emerging prospects for utilizing adjunctive medical therapies, such as bromocriptine, to address this challenging condition. Herein, we report the case of a 26-year-old woman who experienced abrupt and asymmetric bilateral breast enlargement commencing in the second month of her pregnancy. Remarkably, this enlargement persisted for an extended duration of 3 years. Despite the absence of prior medical therapy involving bromocriptine or other interventions, the patient ultimately underwent a simple mastectomy coupled with nipple-areola complex reconstruction. Although bromocriptine treatment holds potential benefits, its availability may vary in different healthcare settings. Therefore, the consideration of surgical management as an alternative approach becomes crucial, particularly when bromocriptine is not accessible or proves ineffective. This approach ensures the appropriate management of gestational gigantomastia, with the choice of treatment tailored to the individual patient's needs and resource availability.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11749582/

Record 270

pmcid: PMC12843043

pmid: 41607568

doi: 10.61882/wjps.14.3.101

title: Surgical Management of Gynecomastia in Asian Men - Clinical Experience and Considerations for Different Patient Types.

authors

- Puah IBK

journal: World journal of plastic surgery

journalAbbr: World J Plast Surg

year: 2025

pubdate: 2025

volume: 14

issue: 3

pages: 101-107

abstract: Gynecomastia, the excess growth of glandular breast tissue in males, is a common condition associated with hormonal imbalance or the use of certain drugs. Although medically benign, persistent gynecomastia often causes psychological distress and adversely affects quality of life. Published literature on the evaluation and management of patients seeking surgical treatment for gynecomastia is predominantly from Western populations. Here, we present insights from our practice in a multi-ethnic Asian population, with >550 cases treated at Amaris B. Clinic, Singapore from 2018-2023. We illustrate these insights and principles with a representative selection of 6 patient cases. We discuss specific considerations for optimizing the consultation process and outcomes in Asian patients, with reference to profiles commonly seen in our local setting. These were: adult males, the most encountered patient profile who are typically overweight/obese; young (adolescent) males, typically overweight, had a history of recent significant weight loss, or lean and experiencing gynecomastia due to puberty-related hormonal changes; males with drug-induced gynecomastia, typically athletes/bodybuilders. Our experience highlights several important considerations, including management of the consultation process, the range of patient expectations, and addressing surgical aspects of management to achieve the desired aesthetic outcomes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12843043/

Record 271

pmcid: PMC12178766

pmid: 40546337

doi: 10.1155/crpe/7659991

title: Suspected Ovotesticular Disorders of Sexual Differentiation in a Phenotypic Male With Ambiguous Genitalia, Light Menstrual Flow, and Synchronous Bilateral Dysgerminoma: A Case Report From Ethiopia.

authors

- Siferih M
- Negasa T
- Yifru M
- Sisay A
- Tadele G
- Adem T
- Gebrie M
- Taye W

journal: Case reports in pediatrics

journalAbbr: Case Rep Pediatr

year: 2025

pubdate: 2025

volume: 2025

issue:

pages: 7659991

abstract: Background: Ovotesticular disorder of sexual differentiation (DSD) is one of the rarest congenital conditions affecting gonadal and sexual development, characterized by the coexistence of ovarian and testicular tissue within an individual. This condition often presents with ambiguous genitalia, atypical pubertal development, or unexpected menstrual activity. This case report details a 14-year-old phenotypic male with ambiguous genitalia, cyclic perineal bleeding, and synchronous bilateral dysgerminoma, underscoring the diagnostic complexities and management challenges encountered in resource-constrained settings. Case Presentation: A 14-year-old individual assigned male at birth and raised as a boy presented with progressive abdominal distension, cyclic perineal bleeding, and absent male secondary sexual characteristics. Physical examination revealed ambiguous genitalia, a small phallic structure, a perineal opening with menstrual blood, and no palpable gonads. Hormonal analysis revealed elevated lactate dehydrogenase and gonadotropins, low testosterone levels, and increased estradiol. Imaging revealed an abdominopelvic mass highly suggestive of ovarian malignancy, and vaginal exploration confirmed Müllerian structures. Laparotomy revealed a 16-cm × 18-cm right adnexal mass, and histopathology confirmed dysgerminoma. The patient was lost to follow-up but returned 6 months later with a contralateral (left) adnexal mass, prompting oncologic referral. Conclusion: Ovotesticular DSD with bilateral dysgerminoma is exceedingly rare and poses significant diagnostic and therapeutic challenges. Early diagnosis, multidisciplinary management, and timely oncologic intervention are crucial for optimizing patient outcomes, especially in resource-limited settings. This case underscores the critical need for heightened awareness, improved access to karyotyping, genetic and hormonal assessments, and long-term follow-up for individuals presenting with ambiguous genitalia and atypical pubertal development.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12178766/

Record 272

pmcid: PMC12651315

pmid: 41300992

doi: 10.3390/cancers17223626

title: Suspicion for Sarcoma: Clinical Presentation, Multi-Modality Imaging Evaluation, and Ultrasound Artificial Intelligence-Based Decision Support.

authors

- Mehran NA
- Rooney E
- Shah H
- Gomolin T
- Zeizafoun N
- Williams D
- Margolies LR
- Chen C

journal: Cancers

journalAbbr: Cancers (Basel)

year: 2025

pubdate: 2025 Nov 11

volume: 17

issue: 22

pages:

abstract: Background/Objective : This study aims to better characterize the clinical presentation, histology, and imaging features of breast sarcomas on mammography, ultrasound, and MRI, in addition to analyzing the effectiveness of AI DS in detecting breast sarcomas. Methods : A retrospective review from 2008-2024 yielded 18 patients with histologically proven breast sarcomas with imaging available for review. Mammography was available for 13 lesions, ultrasound for 19 lesions, and MRI for 9 lesions. Imaging features were classified according to the BI-RADS 5th edition lexicon. Images were reviewed by two radiologists, and consensus was obtained regarding imaging features. AI DS was retrospectively applied to the breast masses identified on ultrasound. Data analysis was performed using descriptive statistics. Results : 17 females and 1 male were included in this study. Mammographic findings varied from solitary masses (3/13 [23.1%]), asymmetries (3/13 [23.1%]), architectural distortion (1/13 [7.7%]), skin thickening (3/13 [23.1%]), focal asymmetry with calcifications (1/13 [7.7%]), or no suspicious findings (2/13 [15.4%]). Sonography often revealed masses with an irregular shape (13/16 [81.2%]), non-circumscribed margins (15/16 [93.7%]), hypoechoic echo pattern (10/16 [62.5%]), and vascular flow (12/16 [75%]). MRI showed heterogeneously enhancing masses (6/9 [66.7%]) or isolated skin enhancement (3/9 [33.3%]). AI DS analyzed 16 masses on ultrasound and identified 15 (93.8%) as suspicious. Conclusions : Breast sarcomas had a variable appearance on breast imaging, ranging from a solitary mass to isolated skin findings. Awareness of how breast sarcomas can present across imaging modalities while using AI DS as an aid may help radiologists in making the correct diagnosis of this rare and aggressive disease.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12651315/

Record 273

pmcid: PMC12243210

pmid: 40634836

doi: 10.1186/s12884-024-07071-0

title: Symptoms of pelvic floor dysfunctions during pregnancy and postpartum.

authors

- de Amorim AC
- Roque LC
- Ito LM
- de Oliveira Murer PG
- Sartori MGF

journal: BMC pregnancy and childbirth

journalAbbr: BMC Pregnancy Childbirth

year: 2025

pubdate: 2025 Jul 10

volume: 25

issue: 1

pages: 742

abstract: BACKGROUND: Monitoring Pelvic Floor Dysfunction (PFD) symptoms during pregnancy is crucial to treating and preventing the onset or worsening of dysfunctions in the postpartum period. This study aimed to evaluate PFD symptoms over time in pregnant and postpartum women. METHODOLOGY: This longitudinal follow-up study involved pregnant and postpartum women. PFD symptoms were assessed using an obstetric history questionnaire and the Australian Pelvic Floor Questionnaire (APFQ). Pregnant women were grouped into two periods: up to 28 weeks (Period 1) and from 28 to 40 weeks of gestation (Period 2). Postpartum women were divided into three periods: up to 6 weeks (Period 3), between 7 and 24 weeks (Period 4), and more than 24 weeks postpartum (Period 5). RESULTS: A total of 46 and 44 pregnant women and 65, 53, and 39 postpartum women were analyzed in Periods 1 to 5, respectively. For pregnant women, non-parametric ANOVA revealed a significant difference ($p = 0.02$) in sexual function between Periods 1 (2.1 ± 2.8) and 2 (1.1 ± 1.8). Among postpartum women, urinary function improved significantly across Periods 3 (9.5 ± 5.9), 4 (4.0 ± 5.1), and 5 (5.7 ± 5.3; $p < 0.001$). Sexual function deteriorated significantly across these periods. Spearman's correlation indicated moderate associations between urinary function and pregnancy ($p < 0.001$, $r = 0.4$) and between prolapse and parity ($p = 0.02$, $r = 0.35$) for pregnant women. For postpartum women, moderate correlations were observed between urinary function and parity ($p = 0.02$, $r = 0.3$), bowel function and age ($p = 0.04$, $r=-0.29$), and sexual function and age ($p = 0.049$, $r=-0.3$). CONCLUSION: The study highlights variations in PFD occurrence during pregnancy and the postpartum period, particularly concerning urinary and sexual function.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12243210/

Record 274

pmcid: PMC12306508

pmid: 40734895

doi: 10.7759/cureus.86961

title: Tattoo Pigment in Breast Tissue: A Rare Case of Pigment Deposition in Gynecomastia.

authors

- Cail GM
- Faust TF
- Steadman MB
- Weaver TL

journal: Cureus

journalAbbr: Cureus

year: 2025

pubdate: 2025 Jun

volume: 17

issue: 6

pages: e86961

abstract: Gynecomastia with associated tissue pigmentation is rarely reported in the literature. We report a rare

case of a 39-year-old male with gynecomastia and unexpected pigmentation within breast tissue, presumed to be related to extensive tattoos. This unusual presentation of gynecomastia, combined with visible pigment deposits in breast and lymphatic tissue, introduces a novel intersection between seemingly unrelated findings. Gynecomastia, a benign enlargement of male breast tissue, is typically idiopathic or hormonally driven. In this case, surgery revealed tattoo-like pigmentation within fibroadipose tissue and lymph nodes. Histopathology confirmed the pigment to be consistent with tattoo ink, raising the possibility of migration via lymphatic channels. While tattoo ink complications are well documented in dermatology, this is the first known report of pigment deposition in male breast tissue in the context of gynecomastia. This raises questions about tattoo pigment migration and potential implications in tissue remodeling or pathology.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12306508/

Record 275

pmcid: PMC11983432

pmid: 40213703

doi: 10.3389/fphar.2025.1570661

title: The characteristics of adverse reactions of three anti-prostate cancer drugs based on Vigiaccess database and bibliometric analysis.

authors

- Wang J
- You J
- Huang W
- Yuan C
- Chen J
- Wang F
- Wang W
- Zhang L

journal: Frontiers in pharmacology

journalAbbr: Front Pharmacol

year: 2025

pubdate: 2025

volume: 16

issue:

pages: 1570661

abstract: BACKGROUND: Androgen antagonists, including apalutamide, darolutamide, and enzalutamide, play a crucial role in the treatment of prostate cancer. This research evaluated the adverse drug reactions (ADRs) associated with the use of these androgen antagonists as reported by the World Health Organization (WHO). Additionally, it compared the adverse drug reaction (ADR) profiles of the three drugs to identify which one presents the lowest risk for individualized patient use. METHODS: This study employed a retrospective descriptive analysis design. We collected adverse event reports for three marketed androgen antagonists from WHO-VigiAccess and analyzed them in combination with a bibliometric analysis. We calculated the percentage of adverse reactions reported for each drug to compare the similarities and differences in adverse reactions among the three drugs. RESULTS: A total of 172,020 adverse events (AEs) associated with three androgen antagonists were reported in VigiAccess at the time of this study. Our findings show apalutamide causes the most endocrine disorders. Darolutamide has the highest rate of blood and lymphatic disorders, while enzalutamide causes the most nervous system disorders. The ten most common ADRs identified were fatigue, rash, death, hot flush, diarrhoea, asthenia, nausea, dizziness, arthralgia, and decreased appetite. CONCLUSION: This study utilizes real data from WHO-VigiAccess, which offers valuable insights for clinical

reference. On one hand, we confirm both existing and potential adverse effects associated with androgen antagonists. On the other hand, We analyzed the possible future research directions, thereby supporting the case for more scientific treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11983432/

Record 276

pmcid: PMC11810079

pmid: 39935901

doi: 10.1080/19317611.2025.2453909

title: The Effect of Sexuality Education During Pregnancy on Female Sexual Function: A Systematic Review and Meta-Analysis Study.

authors

- Ada G
- Hamlacı Başkaya Y
- İlçioğlu K

journal: International journal of sexual health : official journal of the World Association for Sexual Health

journalAbbr: Int J Sex Health

year: 2025

pubdate: 2025

volume: 37

issue: 1

pages: 116-130

abstract: OBJECTIVE: This meta-analysis aimed to synthesize the effect of sexuality education during pregnancy on female sexual function. Although sexual dysfunction in pregnancy is a common problem, it is often overshadowed by other outcomes, such as pregnancy risks or birth complications. Therefore, understanding the impact of educational interventions on women's sexual health is crucial to improving their overall well-being during this period. METHOD: Studies were searched on the electronic databases Google Scholar, Web of Science, Journal Park, the Cochrane Library, EBSCO Medline, Pubmed, and Scopus without imposing any restrictions on the publication year. This meta-analysis used the Preferred Reporting Items for Systematic Reviews and meta-analysis statement guidelines. Findings were analyzed using fixed- or random-effects models based on 95% confidence intervals, p values, and heterogeneity testing. RESULTS: This systematic review included 4 studies with 337 (training: 169 and control: 168) participants due to the limited number of studies on this topic. Interventions in the four studies using the Female Sexual Function Index showed that sexuality education provided to intervention groups had a statistically significant positive effect on the total female sexual function score compared to control groups ($p < .001$, range: 0.552-0.995). CONCLUSION: This meta-analysis provides insights into the implementation of educational interventions considering their positive impact on female sexual function during pregnancy. Despite the limited number of studies (only 4 studies), the findings suggest that sex education leads to significant improvements in many aspects, such as sexual pleasure, vaginal lubrication, orgasm, sexual desire, and overall satisfaction. Furthermore, pregnancy provides an ideal opportunity to normalize and integrate this education into pregnancy care, as it is a time when women often attend antenatal checkups or childbirth classes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11810079/

Record 277

pmcid: PMC12503616

pmid: 41064273

doi: 10.5455/medarh.2025.79.267-270

title: The Main Blood Supply for The Nipple-Areola Complex of Breast Hypertrophy Evaluated by Multi-Slice Computed Tomography.

authors

- Thuy TTH
- Son TT
- Dung PTV
- Nhung LH
- Huong LT
- Nghia PT

journal: Medical archives (Sarajevo, Bosnia and Herzegovina)

journalAbbr: Med Arch

year: 2025

pubdate: 2025

volume: 79

issue: 4

pages: 267-270

abstract: BACKGROUND: Breast hypertrophy is a condition of increased breast volume, occurring at different ages, causing many psychological and physiological problems such as back pain, inflammation of the inframammary folds, and psychological stress1. OBJECTIVE: This study aimed to identify the main arterial sources of blood supply to the nipple-areola complex (NAC), which may apply to extensive hypertrophic breast reduction surgeries. METHODS: Describe a cross-sectional, uncontrolled clinical case series to evaluate the most accurate blood supply to the NAC based on angiography for all patients diagnosed with excessive breast enlargement requiring surgery. RESULTS: 30 female patients were diagnosed with hypertrophic breasts with a minimum volume of 1005cc and a maximum of 2412 cc. Multi-slice computed tomography (MSCT) was indicated preoperatively to determine the blood supplies to the breast; the internal mammary artery (IMA) predominates in 60/60 breasts (100%), and the lateral thoracic artery predominates in 21/60 breasts (35%). Based on the MSCT scan results, the axial flap design carrying the NAC complex has a dominant branch entirely from the IMA: superior medial pedicle in 48/60 breasts (80%), superior pedicle in 11/60 breasts (18.33%) and medial pedicle in 1/60 breasts (1.67%). None of the breasts showed signs of nipple-areola necrosis after surgery, and only two cases (3.3%) of partial NAC necrosis occurred, both in superior flaps. CONCLUSION: Preoperative breast MSCT imaging helps select the type of pedicle that supplies blood to the NAC and minimizes the risk of NAC necrosis during breast reduction surgery in cases of significant hypertrophy and severe grades of ptosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12503616/

Record 278

pmcid: PMC12156052

pmid: 40507620

doi: 10.3390/jcm14113863

title: The Rise and Refinement of Breast Thread Lifting: A Contemporary Review.

authors

- Bogdan RG
- Helgiu A
- Bloanca VA
- Ichim C

- Todor SB
- Iliescu-Glaja M
- Domnariu HP
- Leonte E
- Crainiceanu ZP
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journal: Journal of clinical medicine

journalAbbr: J Clin Med

year: 2025

pubdate: 2025 May 30

volume: 14

issue: 11

pages:

abstract: Breast thread lifting is a minimally invasive technique for correcting mild to moderate ptosis, offering aesthetic enhancement with reduced morbidity compared to traditional mastopexy. This review examines the anatomical underpinnings, clinical indications, technical nuances and limitations of breast thread lifting. The breast's fascial architecture, particularly the role of Cooper's ligaments and the retromammary space, critically influences thread trajectory and vector planning. Classification systems assist in proper patient selection, highlighting the suitability of thread lifts for Grades I-II ptosis with minimal skin excess. Advances in ultrasonography have improved preoperative planning, thread placement accuracy and postoperative monitoring. Various thread types, including PDO, PLLA, PCL and Silhouette Soft, offer different lifting capacities and collagen-stimulatory properties, necessitating tailored material selection. Although thread lifts offer immediate improvements, their transient nature necessitates careful patient counseling to manage expectations regarding durability and potential maintenance sessions. Innovative techniques, including clavicular anchoring and multi-level subdermal scaffolding, have expanded the procedural repertoire. Despite certain limitations, breast thread lifting remains a valuable tool within the aesthetic surgeon's armamentarium, particularly for patients seeking minimally invasive options with shortened recovery periods and favorable psychosocial outcomes. Future developments are expected to further enhance safety, reproducibility and long-term results.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12156052/

Record 279

pmcid: PMC12240139

pmid: 40464157

doi: 10.1097/MNM.0000000000001997

title: The safety and clinical impact of ultra-low-dose FDG-PET imaging in pregnancy-associated breast cancer: the experience of a major tertiary oncology referral centre in the UK and suggested imaging protocol.

authors

- Desouza K
- Fessas P
- Spurrell E
- Papadimitraki E
- Raja F
- Roylance R
- Ottaviani D
- Franks J

- Betal D
- Babar M
- Athanasiou I
- Cariati M
- Patani N
- Hadad S
- Kayani Y
- Walls D
- Scott C
- Rega M
- Arshad M
- Bomanji J
- Vöö S

journal: Nuclear medicine communications

journalAbbr: Nucl Med Commun

year: 2025

pubdate: 2025 Aug 1

volume: 46

issue: 8

pages: 772-779

abstract: BACKGROUND: Pregnancy-associated breast cancer (PABC) is a complex condition affecting 1 in 3000 pregnancies worldwide. While clinical management has improved, the optimal staging approach for PABC remains uncertain. 18 F-fluorodeoxyglucose PET (FDG-PET) imaging is a standard diagnostic tool for many cancers. However, its use in PABC staging is controversial due to potential radiation risks to the foetus. METHODS: This retrospective case series analysed clinical data from six patients with high-risk PABC who underwent FDG-PET imaging for staging between 2022 and 2023. FDG-PET was based on locally implemented ultra-low-dose imaging protocols. The radiation doses to the foetus were dosimetrically estimated based on dose-per-unit activity values and correlated with postpartum neonatal outcomes. RESULTS: The median foetal radiation dose was 0.975 mGy (range 0.6-1.5 mGy) and was below the threshold for deterministic toxicities. PET imaging upstaged nodal involvement in 33% of patients and influenced treatment decisions. FDG-PET imaging provided valuable staging information in all cases. No adverse foetal effects were observed. CONCLUSION: Ultra-low-dose FDG-PET imaging is a valuable tool providing accurate staging information to guide treatment decisions. The low radiation dose associated with this technique makes it a clinically acceptable modality for cancer staging in pregnant women. A larger case series is needed to precisely quantify foetal radiation doses and assess long-term safety.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12240139/

Record 280

pmcid: PMC12538864

pmid: 41120956

doi: 10.1186/s12887-025-06222-2

title: The value of urinary gonadotropin after triptorelin stimulation tests for diagnosing central precocious puberty in girls with overweight or obesity: a single-center study.

authors

- Zeng B

- Zhou Y
- Jia R
- Kong FS
- Huang P
- Chen M
- Ren F
- Xu Z
- Ma Y

journal: BMC pediatrics

journalAbbr: BMC Pediatr

year: 2025

pubdate: 2025 Oct 21

volume: 25

issue: 1

pages: 830

abstract: BACKGROUND: There are different opinions about the effect of body mass index on gonadotropin-releasing hormone stimulation tests in the laboratory diagnosis of precocious puberty in girls. The blood collection of precocious puberty girls with overweight or obesity is more difficult than those with normal body mass index. It is urgent to find non-invasive diagnostic methods for these girls. The purpose of the study was to evaluate the value of urinary gonadotropin after triptorelin stimulation tests for diagnosing central precocious puberty (CPP) in girls with overweight or obesity. METHODS: This was a hospital data-based retrospective study. Overweight or obese girls with precocious puberty who presented to our pediatric endocrine clinic were involved. Triptorelin (Decapeptyl, 0.1 mg) stimulation tests were performed at 8:30 am. On the triptorelin test day, urine sample collected from 08:30 am to 8:30 pm was defined as the first stimulated urine. Serum and urinary luteinizing hormone (LH) were measured by immunochemiluminometric assay. Urinary creatinine (Cr) was also measured. RESULTS: There were 52 girls with overweight or obesity. They were divided into CPP (n = 36) and peripheral precocious puberty (PPP) (n = 16) groups. The stimulated urinary LH (ULH) concentration corrected by Cr (ULH/Cr), ULH concentration, and ULH content in the CPP group were significantly higher than those in the PPP group. When the first stimulated ULH/Cr, ULH concentration and ULH content were ≥ 239.142 IU/mol, 1.690 IU/L and 619.750 IU, and sensitivity for diagnosing CPP in girls with overweight or obesity were 90.9%, 78.8% and 81.8%, respectively, and specificity were all 100%, and the area under the curve were 0.939, 0.932 and 0.909, respectively. CONCLUSIONS: For overweight or obese girls with precocious puberty, the first stimulated ULH/Cr (cut-off value 240 IU/mol) may be used as a possible alternative to the invasive blood detection of triptorelin stimulation tests for diagnosing CPP. TRIAL REGISTRATION: Not applicable.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12538864/

Record 281

pmcid: PMC12716080

pmid: 41426243

doi: 10.2147/JIR.S546797

title: Therapeutic Effect of Brain Awakening and Mind Opening Acupuncture Combined with Simiao Pill on Mice with Benign Prostatic Hyperplasia.

authors

- Luo Z
- Yang X
- Zhang J

- Zhang B
- Zhao Q
- Cui L
- Bo H

journal: Journal of inflammation research

journalAbbr: J Inflamm Res

year: 2025

pubdate: 2025

volume: 18

issue:

pages: 17663-17675

abstract: PURPOSE: Benign prostatic hyperplasia (BPH) is a common condition in older men, but treatment is often associated with complications and high recurrence rates. Previous clinical studies have shown that Brain Awakening and Mind Opening (BAMO) acupuncture and Simiao Pill can improve lower urinary tract symptoms in patients with BPH. This study explores the potential mechanisms behind these treatment methods for BPH using a mice model. METHODS: Six-week-old ICR male mice were divided into 5 groups (n=10). The Mice were injected subcutaneously with TP (5mg/kg/d) / corn oil (equal volume) for 28 days to establish a BPH model and a control group. After successful modelling, the mice were treated with distilled water for 21 days, while the BAMO acupuncture group, the Simiao pill group, and the mixed group were treated accordingly (BAMO acupuncture at Neiguan (PC6), Sanyinjiao (SP6), and Guanyuan (CV4) acupoints for 30 minutes every other day, or Simiao pill once a day). RESULTS: BAMO acupuncture, Simiao pills, and their combination effectively reduced prostate hyperplasia in mice. The observed therapeutic effects likely involve multiple mechanisms: suppression of dihydrotestosterone (DHT) synthesis via reduced testosterone levels and downregulated SRD5A2 expression ($P<0.01$), promotion of prostate cell apoptosis (indicated by an increased BAX/BCL-2 ratio, $P<0.05$), and mitigation of inflammation through decreased levels of pro-inflammatory cytokines IL-6 and TNF-α ($P<0.05$). CONCLUSION: BAMO acupuncture and Simiao pill can restore normal prostate tissue structure in mice through multiple mechanisms: (1) reducing DHT production; (2) promoting apoptosis; (3) reducing inflammatory responses. Future studies could further investigate the hormonal regulation and apoptosis of BAMO acupuncture in mice.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12716080/

Record 282

pmcid: PMC12379338

pmid: 40859190

doi: 10.1186/s12887-025-06076-8

title: Therapeutic outcomes of GnRHa and rhGH in idiopathic central precocious puberty: a focus on psychological and behavioral improvements.

authors

- Chen J
- Cao L
- Chen X
- Zhou X
- Yu Q
- Yang W
- Chen H

journal: BMC pediatrics

journalAbbr: BMC Pediatr

year: 2025

pubdate: 2025 Aug 26

volume: 25

issue: 1

pages: 655

abstract: OBJECTIVE: The psychological issues associated with idiopathic central precocious puberty (ICPP) in girls remain a contentious topic, particularly regarding the necessity and effectiveness of psychological interventions and pharmacological treatments such as gonadotropin-releasing hormone analog (GnRHa) or GnRHa combined with recombinant human growth hormone (rhGH). METHODS: This study aimed to assess the psychological profiles of 95 ICPP girls and their parents, comparing them with 70 healthy controls, and to investigate the potential therapeutic effects of pharmacological interventions. For parents, psychological evaluations were conducted using the Child Behavior Checklist, State Anxiety Inventory, and Positive and Negative Affect Schedule. Whereas for ICPP and control girls, the Children's Depression Inventory and Social Anxiety Scale for Children were administered. RESULTS: Results indicated significant psychological challenges for ICPP girls, including issues with social competence, social withdrawal, defaulters, cruelty, depressive moods, and social anxiety. Correspondingly, their parents exhibited elevated anxiety levels. After 12 months of treatment, the GnRHa group demonstrated notable improvements in aggression, depressive moods, and social anxiety. Meanwhile, improvements were also observed in defaulter behaviors and depressive moods among the GnRHa plus rhGH treated group. Additionally, parental anxiety scores significantly decreased across both treatment groups. CONCLUSION: ICPP girls face a spectrum of psychological difficulties, with their parents also experiencing notable anxiety. Both GnRHa and GnRHa combined with rhGH treatments can reduce parental anxiety and improve the psychological and behavioral problems of ICPP girls. This highlights the importance of addressing psychological health within medical treatment plans for ICPP.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12379338/

Record 283

pmcid: PMC12421314

pmid: 40937165

doi: 10.1002/fsn3.70832

title: Thymol as a Potential Natural Antiemetic: Insights From In Vivo and In Silico Studies.

authors

- Akbor S
- Bappi MH
- Prottay AAS
- Haque F
- Mia N
- Islam T
- Almarhoon ZM
- Gürer ES
- Setzer WN
- Sharifi-Rad J
- Al Hasan S
- Torequl Islam M

journal: Food science & nutrition

journalAbbr: Food Sci Nutr

year: 2025

pubdate: 2025 Sep

volume: 13

issue: 9

pages: e70832

abstract: Nausea and vomiting are common and distressing responses to toxins, chemotherapy, and gastrointestinal disturbances. Although antiemetic medications are available, their adverse effects make safer substitutes necessary. In our study, Thymol (THY), a phenolic monoterpene from essential oils, was evaluated for its antiemetic potential using in vivo and in silico methods. In the in vivo study, emesis was induced in 2-day-old chicks by oral administration of copper sulfate pentahydrate (50 mg/kg). THY was administered orally at doses of 10, 20, and 40 mg/kg, alone or in combination with standard antiemetics ondansetron (ODN), domperidone (DPD), hyoscine butyl bromide (HYS), and promethazine hydrochloride (PRO). The 20 mg/kg dose (THY-20) showed the highest efficacy, significantly ($p < 0.0001$) reducing the number of retches (24.6 ± 2.7; 67.2% reduction) and increasing latency to first retch (52.6 ± 4.2 s; 77.18% increase) compared to the negative control (NC). The THY + ODN combination further enhanced effects (68.53% retch reduction). Molecular docking showed strong binding of THY to 5-HT 3A (-6.4 kcal/mol), D 2 (-7.1 kcal/mol), M 3 (-6.2 kcal/mol), and H 1 (-7.1 kcal/mol) receptors, comparable to standard drugs. Absorption, distribution, metabolism, excretion, and toxicity (ADMET) profiling revealed THY's compliance with Lipinski's Rule, high gastrointestinal absorption, blood-brain barrier permeability, and low toxicity risk. The multi-target binding profile and synergistic potential of THY suggest its promise as a natural, broad-spectrum antiemetic. Further receptor-specific studies and trials in chemotherapy-induced emesis models are recommended to validate its clinical potential.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12421314/

Record 284

pmcid: PMC11730096

pmid: 39713876

doi: 10.4274/jcrpe.galenos.2024.2024-6-26-S

title: Treatment and Follow-up of Congenital Adrenal Hyperplasia Due to 21-hydroxylase Deficiency in Childhood and Adolescence.

authors

- Peltek Kendirci HN
- Ünal E
- Dündar İ
- Buluş AD
- Odabaşı Güneş S
- Şıklar Z

journal: Journal of clinical research in pediatric endocrinology

journalAbbr: J Clin Res Pediatr Endocrinol

year: 2025

pubdate: 2025 Jan 10

volume: 17

issue: Suppl 1

pages: 12-22

abstract: Congenital adrenal hyperplasia (CAH) is an autosomal recessive disease caused by the deficiency of one of the enzymes involved in cortisol synthesis. More than 95% of the cases occur as a result of defects in the

gene encoding 21-hydroxylase (CYP21A2). 21-hydroxylase deficiency has been divided into classical and non-classical forms. In the treatment of classical CAH, it is necessary to replace both glucocorticoid (GC) and mineralocorticoid hormones to prevent salt wasting crisis and reduce excessive corticotropin. In addition to biochemical measurements to evaluate the adequacy of GC and mineralocorticoid treatment; growth rate, body weight, blood pressure and physical examination should be evaluated regularly. There is insufficient data regarding the use of continuous slow-release or modified-release hydrocortisone (HC) preparations and continuous subcutaneous HC infusion, additional/alternative treatment approaches, and cell-based therapies and gene editing technology in children with CAH. GC therapy is recommended in children with inappropriately early onset and rapidly progressing pubarche or accelerated bone age progression, and in adolescents with non-classical CAH (NCCAH) who have overt virilization. In patients with NCCAH, stress doses of HC is recommended for major surgery, trauma, or childbirth but only if the patient has a suboptimal cortisol response to the adrenocorticotropic hormone test. Here, members of the ‘Adrenal Working Group’ of ‘The Turkish Society for Pediatric Endocrinology and Diabetes’ present an evidence-based review with good practice points and recommendations for optimize treatment, and follow-up of children with CAH due to 21-hydroxylase deficiency in the light of the most recent evidence.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11730096/

Record 285

pmcid: PMC12402341

pmid: 40890214

doi: 10.1038/s41598-025-16734-7

title: Triptorelin associated adverse events evaluated using FAERS pharmacovigilance data.

authors

- Jia W
- Wang T

journal: Scientific reports

journalAbbr: Sci Rep

year: 2025

pubdate: 2025 Sep 1

volume: 15

issue: 1

pages: 32042

abstract: Triptorelin, a gonadotropin-releasing hormone(GnRH) agonist, is approved by the US Food and Drug Administration(FDA) for treating advanced prostate cancer, endometriosis, and central precocious puberty(CPP) in children aged ≥ 2 years. This study aimed to characterize the real-world adverse event(AE) profile associated with triptorelin using data from the FDA Adverse Event Reporting System(FAERS). We conducted a retrospective pharmacovigilance study utilizing FAERS reports from the first quarter of 2004 to the third quarter of 2024 (2004Q1-2024Q3). Disproportionality analysis employing four distinct algorithms (Reporting Odds Ratio [ROR], Proportional Reporting Ratio [PRR], Bayesian Confidence Propagation Neural Network [BCPNN], and Multi-item Gamma Poisson Shrinker [MGPS]) was performed to identify potential statistical signals of triptorelin-associated AEs. Among 18,541,994 eligible FAERS reports, 4018 primary suspect reports involving triptorelin were identified. Disproportionality analysis revealed 102 statistically significant Preferred Terms(PTs). Unexpected statistical signals warranting further investigation included defiant behavior and Alzheimer's dementia. The median time-to-onset (TTO) of AEs was 132 days (interquartile range[IQR] 36-361 days). AE reporting exhibited a bimodal distribution, with the highest proportions occurring within the first month (22.59%) and after more than one year (25.07%) following administration. Distinct statistical signal profiles were observed between genders. Analysis of FAERS data elicited statistical signals for both expected and unexpected AEs associated with triptorelin. These findings highlight potential safety signals, particularly defiant behavior and Alzheimer's dementia, which meet the European Medicines Agency (EMA) 2024 criteria for safety signals

requiring further investigation. Continuous pharmacovigilance monitoring is recommended. It is crucial to emphasize that these findings represent statistical associations identified through disproportionality analysis; they require clinical validation and do not establish causality.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12402341/

Record 286

pmcid: PMC11971030

pmid: 40190899

doi: 10.7759/cureus.80107

title: Turner Syndrome With Central Precocious Puberty During Growth Hormone Therapy: Combined Treatment With a Gonadotropin-Releasing Hormone Analog.

authors

- Tanaka K
- Tagaya M

journal: Cureus

journalAbbr: Cureus

year: 2025

pubdate: 2025 Mar

volume: 17

issue: 3

pages: e80107

abstract: Turner syndrome (TS) is a chromosomal disorder characterized by short stature and gonadal dysgenesis. However, precocious puberty is rarely observed in TS, and its optimal management remains unclear. We report a case of a 10-year-old girl with TS and a 45,X/46,XX mosaic karyotype who presented with menarche. She had been receiving growth hormone (GH) therapy since the age of four years and six months. Laboratory and imaging findings confirmed central precocious puberty (CPP), and gonadotropin-releasing hormone analog (GnRHa) therapy was introduced alongside GH therapy. Treatment with GnRHa continued until 11 years and 11 months, while GH therapy was maintained until 14 years and 11 months. The patient ultimately achieved a final height of 140.0 cm (-3.3 SD), within the target height range. This case highlights the rare occurrence of CPP in TS and demonstrates the potential impact of combined GH and GnRHa therapy on growth outcomes. Further studies are needed to establish the optimal treatment strategies, including the appropriate timing and duration of therapy for TS patients with CPP.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11971030/

Record 287

pmcid: PMC12836117

pmid: 41608463

doi: 10.1093/asjof/ojaf148

title: U-Shaped Triple Lipodermal Flap: A Technical Refinement Aimed at Mitigating T-Junction Necrosis in Reduction Mammoplasty.

authors

- Elangovan A
- Natasha
- Shyaka I
- Ramasamy K

journal: Aesthetic surgery journal. Open forum

journalAbbr: Aesthet Surg J Open Forum

year: 2025

pubdate: 2025

volume: 7

issue:

pages: ojaf148

abstract: BACKGROUND: T intersection breakdowns are a difficult problem encountered in Wise pattern breast reductions. OBJECTIVES: This preliminary study aimed to evaluate the efficacy of the U-shaped triple lipo-dermal flap technique in reducing T junction necrosis in superomedial pedicle reduction mammoplasty limited to a fairly low risk group of patients. METHODS: Our prospective cohort study comprised 20 women who underwent breast reduction surgery between May 2023 and May 2024. All the patients who underwent breast reduction surgery for benign breast enlargement met the inclusion criteria. We excluded patients who were smokers, had autoimmune diseases, had a body mass index (BMI) of greater than 30, had diabetes, hypertension, were scheduled for onco-reconstruction or had complication other than wound healing. Data on BMI, ptosis severity, weight of the removed breast tissue, healing rates and follow-up duration were gathered. All patients were operated by a single surgeon. These patients were followed up on postoperative days 7, 10, 14, 21, 28, 40, and monthly. During follow-up complications with regard to wound healing, the scar quality and patient satisfaction were considered. RESULTS: One patient (5%) had experienced epidermolysis and subsequent full thickness dehiscence at the T zone. CONCLUSIONS: The principle behind this strategy is that the incorporation of lipodermal flaps distributes the tension across the suture line, thereby reducing the risk of ischemia at the T junction. The technique is safe, versatile, and easy to execute. It provides a tension-free zone and acting as internal dermal sling. The use of the U-shaped triple lipo-dermal flaps in Wise pattern superior medial pedicle breast reduction demonstrated a 5% rate of wound breakdown at the T junction in a low-risk group of patients. Further studies need to be conducted on a broader breast reduction population is needed to more fully assess the efficacy of this technique.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12836117/

Record 288

pmcid: PMC11905386

pmid: 40093774

doi: 10.17161/kjm.vol18.22874

title: Uncommon Presentation of Metastatic Melanoma to the Breast.

authors

- Veach JA
- Hill M
- Huppe AI

journal: Kansas journal of medicine

journalAbbr: Kans J Med

year: 2025

pubdate: 2025 Jan-Feb

volume: 18

issue: 1

pages: 19-20

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11905386/

Record 289

pmcid: PMC12417314

pmid: 40933294

doi: 10.1002/ccr3.70880

title: Unilateral Gynecomastia and Primary Hypogonadism Following Brucellosis Infection: A Rare Case Report.

authors

- Safdari Y

journal: Clinical case reports

journalAbbr: Clin Case Rep

year: 2025

pubdate: 2025 Sep

volume: 13

issue: 9

pages: e70880

abstract: Brucellosis is a bacterial infection with a wide spectrum of clinical manifestations, yet endocrine complications such as primary hypogonadism and gynecomastia are exceedingly rare. We present the case of a 60-year-old male who was treated for brucellosis and, several weeks after discharge, developed unilateral enlargement of the left breast. Laboratory evaluation revealed significantly elevated gonadotropins (FSH and LH) with markedly decreased serum testosterone levels, indicating primary testicular failure. Sonographic imaging confirmed gynecomastia with a flame-shaped, irregular mass. This case highlights a rare hormonal sequela of brucellosis and emphasizes the importance of endocrine evaluation in patients who present with atypical manifestations following systemic infections.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12417314/

Record 290

pmcid: PMC12828586

pmid: 41585793

doi: 10.3389/fendo.2025.1744280

title: Unveiling the hidden: acquired pediatric hypothyroidism.

authors

- Salerno M
- Capalbo D
- Valenzise M
- Di Mase R
- Morabito LA
- Centonze C
- Wasniewska MG

journal: Frontiers in endocrinology

journalAbbr: Front Endocrinol (Lausanne)

year: 2025

pubdate: 2025

volume: 16

issue:

pages: 1744280

abstract: Acquired hypothyroidism is the most common thyroid disorder in children, with Hashimoto thyroiditis representing the leading cause in iodine-sufficient regions. Children and adolescents may present with a broad spectrum of clinical signs and symptoms, ranging in severity. Unusual or atypical presentations can complicate the differential diagnosis, potentially resulting in delays in both diagnosis and treatment, which may significantly impact growth and developmental outcomes. This review presents four real-life pediatric cases that illustrate both typical and atypical manifestations of acquired hypothyroidism. These cases are accompanied by a comprehensive overview of the condition's etiology, clinical presentation, diagnostic strategies, and management approaches in the pediatric population. The aim of this review is to summarize current knowledge on acquired hypothyroidism in children. By presenting real-world clinical cases, this review highlights key elements that can assist pediatricians in identifying and managing pediatric patients affected by this condition.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12828586/

Record 291

pmcid: PMC12396864

pmid: 40894819

doi: 10.1055/s-0045-1802557

title: Using Ultrasound and Power-Assisted Devices through Lateral Incision: The OCCULT Technique for Gynecomastia-A Multicentric Large Series Study.

authors

- Gupta R
- Bansal P
- Chugh A
- Shah SG
- Chaudhary G

journal: Indian journal of plastic surgery : official publication of the Association of Plastic Surgeons of India

journalAbbr: Indian J Plast Surg

year: 2025

pubdate: 2025 Aug

volume: 58

issue: 4

pages: 285-291

abstract: INTRODUCTION: Gynecomastia is a common condition characterized by male breast enlargement, which can have a profound psychological impact on affected individuals. Surgical intervention is often sought to alleviate these concerns. This study evaluates the lateral port technique for gynecomastia surgery, which combines power-assisted liposuction (PAL) and ultrasound-assisted liposuction (UAL) to achieve optimal surgical outcomes with no scar on front of chest. MATERIALS AND METHODS: A retrospective analysis was conducted on 967 patients who underwent gynecomastia surgery using the Out of Sight, No Cut on Front of Chest Using Ultrasound and Power-Assisted Devices through Lateral Incision (OCCULT) technique between January 2022 and December 2023. The procedures were performed at multiple centers located in New Delhi, Gurugram, and Mumbai, India. This technique involves a single lateral chest wall incision and incorporates both PAL and UAL to ensure effective tissue removal and contouring. Outcomes assessed included surgical efficacy, complication rates, and patient satisfaction. The surgeries were performed by a team of experienced surgeons, ensuring the reproducibility and consistency of the method. RESULTS: The OCCULT technique demonstrated high efficacy and high patient satisfaction. Among the 967 patients, 93.3% reported being satisfied with the

surgical results. Excellent scar quality was reported by 97.1% of the patients. Moreover, the scar was out of sight, that is, no scar on the front of chest was made in the OCCULT technique for gynecomastia surgery. Complications were minimal, with only 0.8% experiencing seroma formation and no cases of necrosis observed. CONCLUSION: The OCCULT technique for gynecomastia surgery is an effective and reproducible method that provides aesthetically favorable results with minimal complications. Its ability to address the psychological and physical burdens of gynecomastia highlights its value in modern surgical practice.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12396864/

Record 292

pmcid: PMC12494393

pmid: 41049522

doi: 10.1297/cpe.2025-0032

title: Van Wyk-Grumbach syndrome: a case report and review of the literature.

authors

- Komatsu Y
- Kikuchi N

journal: Clinical pediatric endocrinology : case reports and clinical investigations : official journal of the Japanese Society for Pediatric Endocrinology

journalAbbr: Clin Pediatr Endocrinol

year: 2025

pubdate: 2025 Oct

volume: 34

issue: 4

pages: 268-274

abstract: Van Wyk-Grumbach syndrome (VWGS) is a rare manifestation of acquired hypothyroidism that was first described in 1960. It is characterized by precocious puberty, delayed bone age, and, in some cases, galactorrhea. We report the case of a 9-yr-old girl with growth retardation, delayed bone age, and vaginal bleeding without pubic hair development. Laboratory tests showed severe hypothyroidism (TSH level: 1,805 μIU/mL; free T4: 0.1 ng/dL), suppressed LH level, elevated estradiol (53 pg/mL) level, and hyperprolactinemia (59.79 ng/mL). Bilateral ovarian cysts were observed. Levothyroxine normalized thyroid function and reduced the number of cysts; however, central puberty progressed within 5 mo, requiring GnRH analog therapy. A review of 44 previously reported female patients revealed consistent findings including delayed bone age, elevated TSH level, suppressed gonadotropin levels, and frequent vaginal bleeding despite the absence of pubic hair. This atypical sequence likely resulted from TSH-induced estradiol secretion without concurrent adrenal androgen activity. This atypical pubertal development sequence may serve as a useful clinical indicator of VWGS. Assessment of thyroid function is warranted in young girls presenting with isolated vaginal bleeding in the absence of pubic hair, not only to avoid misdiagnosis, but more importantly, to ensure the timely initiation of appropriate treatment for underlying hypothyroidism.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12494393/

Record 293

pmcid: PMC11931462

pmid: 40089882

doi: 10.14341/probl13298

title: [Combination of Klinefelter syndrome and the classic form of congenital dysfunction of the adrenal cortex: clinical observation].

authors

- Volkova NI
- Davidenko IY
- Stavitskaya DP
- Kudinova EV

journal: Problemy endokrinologii

journalAbbr: Probl Endokrinol (Mosk)

year: 2024

pubdate: 2024 Mar 11

volume: 71

issue: 1

pages: 27-31

abstract: Congenital adrenal hyperplasia (CAH) is a defect in one of the enzymes or transport proteins involved in the synthesis of cortisol in the adrenal cortex. Virile form of CAH characterized by cortisol deficiency and hyperandrogenism. Klinefelter syndrome is one of the most frequent chromosomal diseases leading to the development of primary hypogonadism. The manifestation of these two diseases could cause difficulties in diagnosis and medical treatment that lead to adverse consequences and affect the quality of life.A 43-years-old patient consulted a physician complaining about the lack of erections for 4 years, breast enlargement. At the age of 3 years based on experienced growth of pubic hair, decreased level of 17-ketosteroids in the urine and genetic analysis diagnosis of CAH, virile form was suspected. Prednisone 5 mg daily was prescribed. At the age of 5, based on phenotypic features and karyotyping Klinefelter Syndrome (XXY) was diagnosed. At the age of 13, stimulating hormonal chorionic gonadotropin drug with only one course of 10 injections was prescribed. At the age of 18, the patient independently canceled the use of prednisone. Further, he did not receive medication therapy for CAH and Klinefelter syndrome. At the age of 42, adrenal CT revealed formation of the left adrenal gland. According to the results of the hormonal activity examination, high levels of aldosterone and renin were detected. A diagnosis of left adrenal aldosteroma was made and a left-sided adrenalectomy was performed. Histological examination diagnosis of aldosteroma did not confirmed. On physical examination, BMI 30 kg/m2, genoid type of obesity, right testicle isn`t palpated, left testicle is dense, reduced in size. Small penis size. Decreased level of total testosterone, normal level of SHBG, LH and FSH was revealed. Ultrasound of the scrotum organs revealed decrease in the size of the testicles and appendages, a volumetric formation of the right testicle. Thus, diagnosis of CAH, virile form and Klinefelter syndrome, primary hypogonadism, right-sided cryptorchidism was confirmed. Hydrocortisone 30 mg daily was prescribed. Hormone replacement therapy with testosterone preparations was not prescribed until surgical treatment of neoplasm of the right testicle will be performed. On the example of this clinical case, we have demonstrate a combination of two endocrine pathologies and serious mistakes were made in the management of this patient. The management of such patients requires a multidisciplinary approach, which will avoid mistakes and improve the prognosis and quality of life of these patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11931462/

Record 294

pmcid: PMC11056625

pmid: 38685997

doi:

title: 1444-nm Nd:YAG for Laser-Assisted Lipolysis: A Minimally Invasive Technique for the Treatment of Pseudogynecomastia.

authors

- Piccolo D
- Mutlag MH

- Ronconi L
- Fusco I
- Bonan P

journal: Eplasty

journalAbbr: Eplasty

year: 2024

pubdate: 2024

volume: 24

issue:

pages: e17

abstract: BACKGROUND: The options for treating pseudogynecomastia are limited, and there is a growing demand for noninvasive breast fat reduction. We evaluated the efficacy and safety of a laser-assisted lipolysis (LAL) device emitting 1444 nm for the treatment of pseudogynecomastia. METHODS: A total of 9 male patients diagnosed with pseudogynecomastia underwent laser procedure with a Nd:YAG laser emitting at 1444 nm. The goal was to generate adequate heat propagation and thermal action in the dermis as homogeneous as possible to induce fat cell lipolysis. The results were clinically evaluated by comparing pictures at baseline and 4 months later. RESULTS: An independent comparison of baseline and post-treatment pictures by a physician evaluator 4 months after treatment revealed significant improvement in all patients. Three subjects (33%) showed grade 4 improvement; 4 (44%) showed grade 3 improvement, and 2 (22%) showed grade 2 improvement. Breast sizes decreased in all patients 4 months after 1444-nm LAL. All patients experienced an incident-free postoperative period. CONCLUSIONS: The LAL procedure performed in this study using 1444-nm Nd:YAG laser appears to be a very effective/comfortable outpatient solution to the problem for patients. It has proven a safe procedure, and all subjects in the study experienced a significant clinical improvement regarding their pseudogynecomastia, with a decrease in breast volume and visible skin contraction.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11056625/

Record 295

pmcid: PMC11454463

pmid:

doi: 10.1210/jendso/bvae163.1607

title: 7745 Hirsutism in young girls, Beyond PCOS.

authors

- Mishra I
- Baliarsinha AK

journal: Journal of the Endocrine Society

journalAbbr: J Endocr Soc

year: 2024

pubdate: 2024 Oct-Nov

volume: 8

issue: Suppl 1

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11454463/

Record 296

pmcid: PMC11453794

pmid:

doi: 10.1210/jendso/bvae163.1694

title: 7745 Hirsutism in young girls: Beyond PCOS.

authors

- Mishra I
- Baliarsinha AK

journal: Journal of the Endocrine Society

journalAbbr: J Endocr Soc

year: 2024

pubdate: 2024 Oct-Nov

volume: 8

issue: Suppl 1

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11453794/

Record 297

pmcid: PMC11455385

pmid:

doi: 10.1210/jendso/bvae163.1543

title: 7802 A Review of 21 Cases of Kallmann Syndrome.

authors

- Kashima T
- Ishimaru M
- Tanimoto E
- Fukui S
- Nakamura C
- Ujita N
- Miyagi H
- Igarashi M
- Doi H
- Yoshii K
- Naiki Y
- Horikawa R

journal: Journal of the Endocrine Society

journalAbbr: J Endocr Soc

year: 2024

pubdate: 2024 Oct-Nov

volume: 8

issue: Suppl 1

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11455385/

Record 298

pmcid: PMC11453518

pmid:

doi: 10.1210/jendso/bvae163.2221

title: 8273 ACTH-Dependent Cushing's Syndrome due to Metastatic Cervical Cancer: A Case Report.

authors

- Bertolacini da Silva TD
- Bertolacini T
- da Costa Kamura V
- de Figueiredo Buchalla P
- Sibanto Simoes AG
- Glina S
- de Sousa Campos ZM
- Eleutério Rosa IS
- Xavier Santi P

journal: Journal of the Endocrine Society

journalAbbr: J Endocr Soc

year: 2024

pubdate: 2024 Oct-Nov

volume: 8

issue: Suppl 1

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11453518/

Record 299

pmcid: PMC11454732

pmid:

doi: 10.1210/jendso/bvae163.2137

title: 8273 ACTH-Dependent Cushing's Syndrome Due To Metastatic Cervical Cancer: A Case Report.

authors

- Bertolacini da Silva TD
- Bertolacini T
- da Costa Kamura V
- de Figueiredo Buchalla P

- Sibanto Simoes AG
- Glina S
- de Sousa Campos ZM
- Eleutério Rosa IS
- Xavier Santi P

journal: Journal of the Endocrine Society

journalAbbr: J Endocr Soc

year: 2024

pubdate: 2024 Oct-Nov

volume: 8

issue: Suppl 1

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11454732/

Record 300

pmcid: PMC10849441

pmid: 38333323

doi: 10.1097/MS9.0000000000001683

title: A 5 kg giant breast lipoma in a 40-year-old woman: a case report.

authors

- Sharma S
- Rayamajhi A
- Chapagain A
- Shrestha JM

journal: Annals of medicine and surgery (2012)

journalAbbr: Ann Med Surg (Lond)

year: 2024

pubdate: 2024 Feb

volume: 86

issue: 2

pages: 1229-1233

abstract: INTRODUCTION AND IMPORTANCE: Lipomas are benign tumours composed of adipocytes surrounded by a thin fibrous capsule. Although they make up 16% of mesenchymal tumours, the occurrence of breast lipomas is uncertain. Giant lipomas, measuring 10 cm or more in diameter or weighing at least 1000 g, commonly occur in the upper back, neck, and thigh, and are rare in the breast. Given its rarity, accurately diagnosing a giant breast lipoma is crucial to prevent potential overtreatment, as it might otherwise be mistaken for a malignant tumour. CASE PRESENTATION: A 40-year-old woman presented with a painless, gradually enlarging left breast mass. Physical examination and imaging studies revealed a lipoma-compatible mass. Surgery was performed, and the 5 kg mass was enucleated and identified as a lipoma on histopathology. The patient had an uneventful postoperative recovery and was satisfied with the outcome of the surgery after 2 months of follow-up. CLINICAL DISCUSSION: Breast lipomas are more common in the 40-60 years age group, with giant lipomas occurring more frequently in the latter half of this age range. They can mimic various breast

conditions, including neoplastic and non-neoplastic conditions, and are often treated with surgical excision to avoid recurrence. The location of the lipoma in the breast can be subcutaneous or intramuscular, and preserving the future pedicle of reduction mammoplasty/mastopexy is essential. CONCLUSION: Giant breast lipoma is an infrequent condition that can manifests as progressive enlargement of the breast, posing a diagnostic challenge due to its resemblance to various benign or malignant pathologies.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10849441/

Record 301

pmcid: PMC10967001

pmid: 38530023

doi: 10.1177/03000605241232520

title: A case of mild partial androgen insensitivity syndrome in a juvenile boy.

authors

- Wang F
- Shao S
- He W
- Hu S

journal: The Journal of international medical research

journalAbbr: J Int Med Res

year: 2024

pubdate: 2024 Mar

volume: 52

issue: 3

pages: 3000605241232520

abstract: Androgen insensitivity syndrome (AIS) is a rare disorder with X-linked recessive inheritance in 46 XY patients. The clinical manifestations vary between patients, especially regarding external genitalia development. Herein, the case of AIS in a 13-year-old male, who was born with hypospadias and presented to the hospital with gynaecomastia that had developed from 8 years of age, is reported. No micropenis, cryptorchidism or bifid scrotum were found. Testis volume was 12 ml on both sides. His testosterone and luteinizing hormone levels were normal compared with sex- and age-adjusted reference range. His bone age was approximately 13 years according to Greulich-Pyle assessment. Sequence analysis of the androgen receptor (AR) gene revealed a mutation (c.2041A>G) in exon 4, a novel mutation site in the AR gene. Prediction analysis suggested this to be a disease-causing variant. A milder clinical presentation and normal hormone levels in cases of partial AIS might differ from the usually reported signs and symptoms. A diagnosis of AIS should not be ignored in teenage patients who present with gynaecomastia and hypospadias, but normal hormone levels.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10967001/

Record 302

pmcid: PMC11004339

pmid: 38601752

doi: 10.3389/fonc.2024.1359886

title: A case report and a literature review of double mammary pseudoangiomatous stromal hyperplasia associated with galactoma during pregnancy.

authors

- Yu M

- Shi G
- Gao Z
- Wu K
- Wei C
- Li X
- Miao X

journal: Frontiers in oncology

journalAbbr: Front Oncol

year: 2024

pubdate: 2024

volume: 14

issue:

pages: 1359886

abstract: Pseudoangiomatous stromal hyperplasia (PASH) is a benign interstitial hyperplasia of the breast that usually occurs in premenopausal or perimenopausal women. It is usually characterized by localized lesions or clear boundary masses, and diffuse double breast enlargement is rare. PASH is considered a hormone-dependent disease that is commonly progesterone related. There are no imaging characteristics, and both benign and suspicious malignant signs can be seen. The definitive diagnosis of PASH depends on a pathological diagnosis, and it is necessary to be vigilant in distinguishing between benign and malignant tumors with similar breast histopathology. Here, we report the case of a 23-year-old multipara patient with bilateral diffuse pseudoangiomatous stromal hyperplasia of the breast during pregnancy who presented with macromastia and reviewed the literature to further understand the clinical features, pathological diagnosis, differential diagnosis, treatment and prognosis of pseudoangiomatous stromal hyperplasia of the breast.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11004339/

Record 303

pmcid: PMC11184532

pmid: 38894785

doi: 10.7759/cureus.60575

title: A Comprehensive Review on the Role of Melatonin's Anesthetic Applications in Pediatric Care.

authors

- Subhadarshini S
- Taksande K

journal: Cureus

journalAbbr: Cureus

year: 2024

pubdate: 2024 May

volume: 16

issue: 5

pages: e60575

abstract: Anesthesia is critical to pediatric care, ensuring the safety and comfort of children undergoing medical procedures. With a growing interest in alternative anesthetic agents, melatonin has emerged as a promising candidate due to its sedative, analgesic, anti-inflammatory, and neuroprotective properties. This comprehensive review explores the potential applications of melatonin in pediatric anesthesia. We delve into the

pharmacological characteristics of melatonin, its anesthetic properties, and its clinical applications in pediatric care, including preoperative sedation, adjunct to general anesthesia, postoperative pain management, and prevention of emergence delirium. Additionally, we discuss the safety profile of melatonin, potential adverse effects, and comparative analysis with traditional anesthetics. Finally, we highlight future research directions to provide insights into melatonin's role in pediatric anesthesia and its implications for clinical practice.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11184532/

Record 304

pmcid: PMC11431469

pmid: 39330001

doi: 10.3390/curroncol31090373

title: A Non-Randomized Comparison of Online and In-Person Formats of the Canadian Androgen Deprivation Therapy Educational Program: Impacts on Side Effects, Bother, and Self-Efficacy.

authors

- Walker LM
- Sears CS
- Wibowo E
- Robinson JW
- Matthew AG
- McLeod DL
- Wassersug RJ

journal: Current oncology (Toronto, Ont.)

journalAbbr: Curr Oncol

year: 2024

pubdate: 2024 Aug 28

volume: 31

issue: 9

pages: 5040-5056

abstract: Although Androgen Deprivation Therapy (ADT) is effective in controlling prostate cancer (PCa) and increasing survival, it is associated with a myriad of side effects that cause significant morbidity. Previous research has shown that PCa patients starting on ADT are neither fully informed nor well-equipped to manage the breadth of ADT's side effects. The ADT Educational Program (a 1.5 h interactive class plus a book) was developed as an evidence-based resource for patients dealing with ADT. Our aim here was to compare the efficacy of an online version of the class with a previously assessed in-person version of the class. Using mixed MANOVAs within a non-randomized comparison design, we assessed: (1) changes in patients' experiences of self-efficacy to manage and bother associated with side effects approximately 10 weeks after attending a class, and (2) potential differences in these variables between online and in-person class formats. Side effect bother decreased from pre- to post-class but did not differ between in-person (n = 94) and online (n = 137) class cohorts. While self-efficacy to manage side effects was slightly higher post-class in both cohorts, the increase was not statistically significant. Average self-efficacy ratings were significantly higher among in-person versus online class participants ($p < 0.05$; $\eta p 2 = 0.128$). Both online and in-person classes are associated with a significant reduction in the severity of side effect bother reported by PCa patients, suggesting non-inferiority of online versus in-person formats. Online classes offer greater accessibility to the program for patients outside the reach of in-person classes, increasing the availability of the program to more PCa patients and family members across Canada.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11431469/

Record 305

pmcid: PMC10970221

pmid: 38542117

doi: 10.3390/ijms25063143

title: A Novel Mutation in the INSR Gene Causes Severe Insulin Resistance and Rabson-Mendenhall Syndrome in a Paraguayan Patient.

authors

- Rojas Velazquez MN
- Blanco F
- Ayala-Lugo A
- Franco L
- Jolly V
- Di Tore D
- Martínez de Lapiscina I
- Janner M
- Flück CE
- Pandey AV

journal: International journal of molecular sciences

journalAbbr: Int J Mol Sci

year: 2024

pubdate: 2024 Mar 8

volume: 25

issue: 6

pages:

abstract: Rabson-Mendenhall syndrome (RMS) is a rare autosomal recessive disorder characterized by severe insulin resistance, resulting in early-onset diabetes mellitus. We report the first case of RMS in a Paraguayan patient. The patient is a 6-year-old girl who presented with hypertrichosis, acanthosis nigricans, nephrocalcinosis, and elevated levels of glucose and insulin that served as diagnostic indicators for RMS. Genetic testing by next-generation sequencing (NGS) revealed two pathogenic variants in exons 2 and 19 of the INSR gene: c.332G>T (p.Gly111Val) and c.3485C>T (p.Ala1162Val), in combined heterozygosis. The novel INSR c. 332G>T variant leads to the substitution of glycine to valine at position 111 in the protein, and multiple in silico software programs predicted it as pathogenic. The c.3485C>T variant leads to the substitution of alanine to valine at position 1162 in the protein previously described for insulin resistance and RMS. The management of RMS is particularly challenging in children, and the use of metformin is often limited by its side effects. The patient was managed with nutritional measures due to the early age of onset. This report expands the knowledge of RMS to the Paraguayan population and adds a novel pathogenic variant to the existing literature.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10970221/

Record 306

pmcid: PMC11772895

pmid: 39877787

doi: 10.7759/cureus.76546

title: A Rare Case of Juvenile Granulosa Cell Tumor in a Premenarchal Female: Clinical Presentation, Diagnosis, and Management.

authors

- Kanna E
- Sfetsiori A
- Batsari E
- Lamprinou Z
- Skondras I

journal: Cureus

journalAbbr: Cureus

year: 2024

pubdate: 2024 Dec

volume: 16

issue: 12

pages: e76546

abstract: Juvenile granulosa cell tumors (JGCTs), a rare type of ovarian tumor, are predominantly seen in premenarchal girls. We report a case of a 4.5-year-old girl with precocious puberty and a left ovarian JGCT, confirmed through imaging and histopathology. The patient underwent a fertility-sparing unilateral salpingo-oophorectomy, and no recurrence was observed after one year of follow-up. Surgery is the mainstay of treatment for early-stage JGCTs, and the role of adjuvant chemotherapy in advanced cases remains unclear. This report underscores the importance of early diagnosis and long-term monitoring in pediatric patients with ovarian masses.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11772895/

Record 307

pmcid: PMC10962068

pmid: 38523311

doi: 10.1186/s13256-024-04438-x

title: A rare variant of mullerian agenesis: a case report and review of the literature.

authors

- Tayade SD
- Mehdi N
- Dube R
- Bose V
- Ameer A
- Hakim ZT
- Wattiez A

journal: Journal of medical case reports

journalAbbr: J Med Case Rep

year: 2024

pubdate: 2024 Mar 25

volume: 18

issue: 1

pages: 126

abstract: INTRODUCTION: Menstruation is a developmental milestone and usually marks healthy and normal pubertal changes in females. Menarche refers to the onset of first menstruation in a female. The causes of primary amenorrhea include outflow tract abnormalities, resistant endometrium, primary ovarian insufficiency, and disorders of the hypothalamus, pituitary, or other endocrine glands. A rare variant of mullerian agenesis, which warrants an individualized approach to management, is presented here. CASE REPORT: We present here the case of a 25-year-old Indian female with pain in the lower abdomen and primary amenorrhea. After a thorough history, clinical examination, imaging, and diagnostic laparoscopy, two small uteri, a blind upper half vagina, bilateral polycystic ovaries, and a blind transverse connection between the two uteri-a horseshoe band cervix-were detected, which confirmed the diagnosis of mullerian agenesis. There was evidence of adenomyosis in the mullerian duct element. This is a rare form of Müllerian abnormality with an unusual presentation. CONCLUSION: Mullerian agenesis is the most common cause of primary amenorrhea with well-developed secondary sexual characteristics. There are various forms of mullerian agenesis. Most of the cases are managed by a multidisciplinary team. Rare variants warrant an individualized approach to management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10962068/

Record 308

pmcid: PMC11242241

pmid: 38999547

doi: 10.3390/jcm13133983

title: A Real-World Safety Profile in Neurological, Skin, and Sexual Disorders of Anti-Seizure Medications Using the Pharmacovigilance Database of the Korea Adverse Event Reporting System (KAERS).

authors

- Kim D
- Lee S

journal: Journal of clinical medicine

journalAbbr: J Clin Med

year: 2024

pubdate: 2024 Jul 8

volume: 13

issue: 13

pages:

abstract: (1) Background : The utilization of high-quality evidence regarding the safety of anti-seizure medications (ASMs) is constrained by the absence of standardized reporting. This study aims to examine the safety profile of ASMs using real-world data. (2) Methods : The data were collected from the Korea Adverse Event Reporting System Database (KAERS-DB) between 2012 and 2021. In total, 46,963 adverse drug reaction (ADR)-drug pairs were analyzed. (3) Results : At the system organ class level, the most frequently reported classes for sodium channel blockers (SCBs) were skin (37.9%), neurological (16.7%), and psychiatric disorders (9.7%). For non-SCBs, these were neurological (31.2%), gastrointestinal (22.0%), and psychiatric disorders (18.2%). The most common ADRs induced by SCBs were rash (17.8%), pruritus (8.2%), and dizziness (6.7%). Non-SCBs were associated with dizziness (23.7%), somnolence (13.0%), and nausea (6.3%). Rash, pruritus, and urticaria occurred, on average, two days later with SCBs compared to non-SCBs. Sexual/reproductive disorders were reported at a frequency of 0.23%. SCBs were reported as the cause more frequently than non-SCBs (59.8% vs. 40.2%, Fisher's exact test, $p < 0.0001$). (4) Conclusions : Based on real-world data, the safety profiles of ASMs were identified. The ADRs induced by SCBs exhibited different patterns when compared to those induced by non-SCBs.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11242241/

Record 309

pmcid: PMC11204990

pmid: 38933431

doi: 10.11604/pamj.2024.47.142.39746

title: A review of epidemiology of lymphatic filariasis in Nigeria.

authors

- Waje T
- Iliyasu C
- Yaki LM
- Auta IK

journal: The Pan African medical journal

journalAbbr: Pan Afr Med J

year: 2024

pubdate: 2024

volume: 47

issue:

pages: 142

abstract: Lymphatic filariasis is a neglected tropical disease that affects the lymphatic system of humans. The major etiologic agent is a nematode called Wuchereria bancrofti, but Brugia malayi and Brugia timoriare sometimes encountered as causative agents. Mosquitoes are the vectors while humans the definitive hosts respectively. The burden of the disease is heavier in Nigeria than in other endemic countries in Africa. This occurs with increasing morbidity and mortality at different locations within the country, the World Health Organization recommended treatments for lymphatic filariasis include the use of Albendazole (400mg) twice per year in co-endemic areas with loa loa, Ivermectin (200mcg/kg) in combination with Albendazole (400mg) in areas that are co-endemic with onchocerciasis, ivermectin (200mcg/kg) with diethylcarbamazine citrate (DEC) (6mg/kg) and albendazole (400mg) in areas without onchocerciasis. This paper covered a systematic review, meta-analysis, and scoping review on lymphatic filariasis in the respective geopolitical zones within the country. The literature used was obtained through online search engines including PubMed and Google Scholar with the heading "lymphatic filariasis in the name of the state", Nigeria. This review revealed an overall prevalence of 11.18% with regional spread of Northwest (1.59%), North Central and North East, (4.52%), South West (1.26%), and South-South with South East (3.81%) prevalence. The disease has been successfully eliminated in Argungu local government areas (LGAs) of Kebbi State, Plateau, and Nasarawa States respectively. Most clinical manifestations (31.12%) include hydrocele, lymphedema, elephantiasis, hernia, and dermatitis. Night blood samples are appropriate for microfilaria investigation. Sustained MDAs, the right testing methods, early treatment of infected cases, and vector control are useful for the elimination of lymphatic filariasis for morbidity management and disability prevention in the country. Regional control strategies, improved quality monitoring of surveys and intervention programs with proper records of morbidity and disability requiring intervention are important approaches for the timely elimination of the disease in Nigeria.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11204990/

Record 310

pmcid: PMC11431527

pmid: 39335737

doi: 10.3390/diagnostics14182058

title: A Unique Case of Unilateral Pseudogynecomastia.

authors

- Kountouri I

- Katsarelas I
- Kokkali E
- Giotas A
- Gkogkos C
- Chatzinas D
- Nachopoulos P
- Faseki A
- Panagiotou A
- Polychronidis A
- Chandolias M
- Gkiatas N
- Manolakaki D
- Dimasis P

journal: Diagnostics (Basel, Switzerland)

journalAbbr: Diagnostics (Basel)

year: 2024

pubdate: 2024 Sep 17

volume: 14

issue: 18

pages:

abstract: BACKGROUND/OBJECTIVES: Gynecomastia is a common condition characterized by the benign enlargement of male breast tissue, often resulting from hormonal imbalances. A rare variant, unilateral pseudogynecomastia, involves enlargement due to adipose tissue accumulation without glandular proliferation and can be associated with occupational factors. METHODS: We report the case of a 45-year-old male mechanic presenting with unilateral enlargement of the left breast. The patient reported daily microtrauma on his left axilla and chest wall. The clinical evaluation and imaging revealed lipomatosis with pronounced fibrous tissue and no glandular tissue involvement. The hormonal assays were within the normal limits. The patient underwent surgical excision of excess adipose tissue using the Kornstein technique, preserving the nipple-areola complex. RESULTS: The histopathological examination confirmed the absence of malignancy. The postoperative recovery was uneventful, and the follow-up examination at 12 months demonstrated a symmetrical breast appearance with no recurrence. This case underscores the importance of differentiating pseudogynecomastia from true gynecomastia and recognizing potential occupational risks. Surgical management using techniques that preserve the nipple-areola complex can achieve excellent cosmetic outcomes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11431527/

Record 311

pmcid: PMC11266290

pmid: 39050538

doi: 10.3389/fmed.2024.1425675

title: Acne treatment: research progress and new perspectives.

authors

- Li Y
- Hu X
- Dong G

- Wang X
- Liu T

journal: Frontiers in medicine

journalAbbr: Front Med (Lausanne)

year: 2024

pubdate: 2024

volume: 11

issue:

pages: 1425675

abstract: Acne is a chronic inflammatory skin disease that primarily affects adolescents and is attributed to various factors, including hormonal changes, genetic predisposition, and environmental influences. It typically manifests in areas rich in sebaceous glands such as the face, chest, and back. Symptoms of acne can range from mild to severe and may present as pimples, pustules, nodules, cysts, and scarring. The appearance of acne can significantly impact both the physical and mental well-being of patients, potentially leading to feelings of anxiety, depression, and social withdrawal. The pathogenesis of acne is multifaceted involving genetic predisposition as well as environmental factors such as hormonal imbalances, inflammation, abnormal follicular sebaceous unit keratinization, proliferation of follicular microorganisms like Propionibacterium acnes , increased sebum production, and dietary influences. Traditional treatment methods for acne include topical drug therapy, oral drug therapy, photoelectric therapy, and chemical peeling. With ongoing research into the pathogenesis of acne, treatment methods are rapidly evolving with novel antibiotics, probiotics, biological agents, topical anti-androgen drugs, topical vitamin A acid metabolism blockers, antimicrobial peptides, immunotherapy, micro-needling, and micro-needling patches. This article aims to provide a comprehensive review of recent advancements in acne treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11266290/

Record 312

pmcid: PMC10853011

pmid: 38333441

doi: 10.7759/cureus.51969

title: An Unusual Case of Gynecomastia Associated With Subclinical Hyperthyroidism.

authors

- Rojas PL
- Ucar R
- Nessa L
- Perez MC
- Suravajjala D

journal: Cureus

journalAbbr: Cureus

year: 2024

pubdate: 2024 Jan

volume: 16

issue: 1

pages: e51969

abstract: Gynecomastia in males is a medical condition that manifests as the abnormal enlargement of male

breast tissue and has a variety of potential causes, which mainly classify as physiologic (infancy, puberty, elderly) and pathologic (hyperthyroidism, medications, cirrhosis, chronic kidney disease (CKD), malignancies). Pathologic causes mainly result from hormonal imbalances. While gynecomastia has been documented in cases of Graves' disease, it is rarely the presenting symptom with very few cases reported in the literature. Here we report an uncommon case of a 65-year-old male with bilateral gynecomastia who presented to his primary care physician (PCP) with concern for breast sensitivity and enlargement. Ultrasound of his breasts showed bilateral findings consistent with gynecomastia. Initial lab demonstrated suppressed thyroid-stimulating hormone (TSH) levels high, normal FT4, elevated estradiol level, elevated sex hormone-binding globulin (SHBG), and elevated total testosterone. The patient was seen by an Endocrinologist six months post-symptom onset and reported that his symptoms had resolved spontaneously.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10853011/

Record 313

pmcid: PMC11766986

pmid: 39859945

doi: 10.3390/life15010005

title: Analysis of Clinically Symptomatic Patients to Differentiate Inflammatory Breast Cancer from Mastitis in Asian Women.

authors

- Mun HS
- Oh HY

journal: Life (Basel, Switzerland)

journalAbbr: Life (Basel)

year: 2024

pubdate: 2024 Dec 24

volume: 15

issue: 1

pages:

abstract: PURPOSE: To differentiate inflammatory breast cancer (IBC) from mastitis in Asian women presenting with symptoms of inflammation. METHODS: Between January 2012 and June 2024, 101 Asian women with symptoms of inflammation underwent breast ultrasound (US). Clinical and demographic data were extracted from patients' medical records. US analysis assessed lesion bilaterality, location, type, size, internal changes, and lymph node status. Patients with suspicious findings had US-guided biopsies, and pathology reports were reviewed for tumor histology and immunohistochemical markers. Logistic regression was used to determine odds ratios. RESULTS: Of the 101 participants, 14 (13.9%) were diagnosed with IBC and 87 (86.1%) were diagnosed with mastitis. Patients with IBC were significantly older (46.4 vs. 38.4 years, $p = 0.020$) and showed a higher prevalence of postmenopausal status (57.1% vs. 12.6%, $p < 0.0001$). These patients experienced a longer symptom onset duration (37.7 vs. 12.7 days, $p = 0.002$) and more frequent localized symptoms like swelling (50.0% vs. 13.8%, $p = 0.004$). US findings showed that 21.4% of IBC lesions involved the entire breast, compared to only 1.1% in patients with mastitis ($p = 0.001$). Biopsy results revealed that invasive ductal carcinoma was the most common malignancy (78.6%). Logistic regression identified symptom onset duration (adjusted odds ratio (OR) 1.07, $p = 0.014$) and swelling (adjusted OR 15.24, $p = 0.016$) as significant predictors of IBC. CONCLUSION: In Asian women, age, menopausal status, symptom onset duration, and swelling are effective in differentiating IBC from mastitis. Logistic regression confirmed that symptom onset duration and swelling are significant predictors of IBC, with US findings indicating larger lesion sizes and more frequent whole-breast involvement.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11766986/

Record 314

pmcid: PMC10889606

pmid: 38398243

doi: 10.3390/jcm13040929

title: Androgen Insensitivity Syndrome with Bilateral Gonadal Sertoli Cell Lesions, Sertoli-Leydig Cell Tumor, and Paratesticular Leiomyoma: A Case Report and First Systematic Literature Review.

authors

- Karseladze AI
- Asaturova AV
- Kiseleva IA
- Badlaeva AS
- Tregubova AV
- Zaretsky AR
- Uvarova EV
- Zanelli M
- Palicelli A

journal: Journal of clinical medicine

journalAbbr: J Clin Med

year: 2024

pubdate: 2024 Feb 6

volume: 13

issue: 4

pages:

abstract: Androgen insensitivity syndrome (AIS) is a rare Mendelian disorder caused by mutations of the androgen receptor (AR) gene on the long arm of the X chromosome. As a result of the mutation, the receptor becomes resistant to androgens, and hence, karyotypically male patients (46,XY) carry a female phenotype. Their cryptorchid gonads are prone to the development of several types of tumors (germ cell, sex cord stromal, and others). Here, we report a 15-year-old female-looking patient with primary amenorrhea who underwent laparoscopic gonadectomy. Histologically, the patient's gonads showed Sertoli cell hamartomas (SCHs) and adenomas (SCAs) with areas of Sertoli-Leydig cell tumors (SLCTs) and a left-sided paratesticular leiomyoma. Rudimentary Fallopian tubes were also present. The patient's karyotype was 46,XY without any evidence of aberrations. Molecular genetic analysis of the left gonad revealed two likely germline mutations-a pathogenic frameshift deletion in the AR gene (c.77delT) and a likely pathogenic missense variant in the RAC1 gene (p.A94V). Strikingly, no somatic mutations, fusions, or copy number variations were found. We also performed the first systematic literature review (PRISMA guidelines; screened databases: PubMed, Scopus, Web of Science; ended on 7 December 2023) of the reported cases of patients with AIS showing benign or malignant Sertoli cell lesions/tumors in their gonads (n = 225; age: 4-84, mean 32 years), including Sertoli cell hyperplasia (1%), Sertoli cell nodules (6%), SCHs (31%), SCAs (36%), Sertoli cell tumors (SCTs) (16%), and SLCTs (4%). The few cases (n = 14, 6%; six SCAs, four SCTs, two SLCTs, and two SCHs) with available follow-up (2-49, mean 17 months) showed no evidence of disease (13/14, 93%) or died of other causes (1/14, 7%) despite the histological diagnosis. Smooth muscle lesions/proliferations were identified in 19 (8%) cases (including clearly reported rudimentary uterine remnants, 3 cases; leiomyomas, 4 cases). Rudimentary Fallopian tube(s) were described in nine (4%) cases. Conclusion: AIS may be associated with sex cord/stromal tumors and, rarely, mesenchymal tumors such as leiomyomas. True malignant sex cord tumors can arise in these patients. Larger series with longer follow-ups are needed to estimate the exact prognostic relevance of tumor histology in AIS.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10889606/

Record 315

pmcid: PMC10839862

pmid: 38318005

doi: 10.1016/j.heliyon.2024.e24413

title: Angiosarcoma of the breast: A review.

authors

- An R
- Men XJ
- Ni XH
- Wang WT
- Wang CL

journal: Heliyon

journalAbbr: Heliyon

year: 2024

pubdate: 2024 Feb 15

volume: 10

issue: 3

pages: e24413

abstract: Breast angiosarcoma is a rare and highly aggressive malignancy with a poor prognosis. It can occur spontaneously or be associated with factors such as radiation therapy or chronic lymphedema. The etiology and pathogenesis of this disease are still unclear, the clinical symptoms and imaging findings lack specificity, and the pathological morphology is diverse, which is easy to be confused with other diseases. There is no clear guideline for surgical treatment. Although the optimal surgical approach remains unclear, the ultimate goal is surgical excision with optimal margins, which remains the primary method of treatment. In clinical practice, the choice of the surgical approach should be made by considering the tumor size, pathological type, and patient preferences. In clinical practice, the selection of surgical methods should be carried out with comprehensive consideration of tumor size, pathological types and patients' wishes. There is no clear consensus on whether radiotherapy and chemotherapy should be carried out after surgery, and its optimal program and efficacy are uncertain. This article reviews the etiology, clinical manifestations, pathological features, imaging findings, treatment, prognosis and other aspects of breast angiosarcoma, so as to strengthen clinicians' overall understanding of this disease and avoid missed diagnosis and misdiagnosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10839862/

Record 316

pmcid: PMC10998210

pmid: 38586368

doi: 10.1016/j.heliyon.2024.e28616

title: Anti-cancer mechanisms of natural isoflavones against melanoma.

authors

- Liang C
- Wang P
- Li M
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- Lai KP
- Chen J

journal: Heliyon

journalAbbr: Heliyon

year: 2024

pubdate: 2024 Apr 15

volume: 10

issue: 7

pages: e28616

abstract: The incidence of skin-related neoplasms has generally increased in recent years. Melanoma arises from malignant mutations in melanocytes in the basal layer of the epidermis and is a fatal skin cancer that seriously threatens human health. Isoflavones are polyphenolic compounds widely present in legumes and have drawn scientists' attention, because they have good efficacy against a variety of cancers, including melanoma, without significant toxic side effects and resistance. In this review article, we summarize the research progress of isoflavones in melanoma, including anti-melanoma roles and mechanisms of isoflavones via inhibition of tyrosinase activity, melanogenesis, melanoma cell growth, invasion of melanoma cells, and induction of apoptosis in melanoma cells. This information is important for the prevention, clinical treatment, and prognosis and survival of melanoma.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10998210/

Record 317

pmcid: PMC10912972

pmid: 38444408

doi: 10.4102/sajpsychiatry.v30i0.2133

title: Antipsychotics-related hyperprolactinaemia among patients with schizophrenia in Maiduguri.

authors

- Shettima FB
- Wakil MA
- Sheikh TL
- Abdulaziz M
- Wakawa IA
- Beida O

journal: The South African journal of psychiatry : SAJP : the journal of the Society of Psychiatrists of South Africa

journalAbbr: S Afr J Psychiatr

year: 2024

pubdate: 2024

volume: 30

issue:

pages: 2133

abstract: BACKGROUND: Hyperprolactinaemia among patients on antipsychotic medications is generally overlooked due to lack of outwardly visible symptoms, patient resistance to reporting because the symptoms are perceived as shameful, or to clinician's insufficient knowledge. AIM: The study aimed to evaluate the patterns and correlates of hyperprolactinemia among patients with schizophrenia on antipsychotic medications. SETTING: The study was conducted in a psychiatric facility in Maiduguri, Northeastern Nigeria. METHODS: A total of 209 patients with schizophrenia were evaluated through a cross-sectional design and assayed for serum

prolactin with ELISA Kits. Frequencies and percentages were tabulated for categorical variables. Variables with significant associations with hyperprolactinaemia on chi-square (p < 0.05) were subjected to logistic regression analysis. RESULTS: The prevalence of hyperprolactinaemia was 45.9% in all patients on antipsychotic medication. The prevalence because of the use of typical and atypical antipsychotics was 51.5% and 25.0%, respectively. Hyperprolactinaemia was significantly associated with typical antipsychotics (β = 0314, p = 0.002), high overall drug dosage (β = 2.340, p = 0.003), high-dose typical antipsychotics (β = 3.228, p = 0.000), twice daily dosing frequency (β = 2.751, p = 0.001) and polypharmacy (β = 1.828, p = 0.0024). CONCLUSION: The findings support that patients on typical, high-dose antipsychotic medications and polypharmacy have a high prevalence of hyperprolactinaemia. As hyperprolactinaemia is often undetectable, screening and patient psycho-education on the significance of the signs and symptoms of hyperprolactinaemia is required for necessary clinical intervention. CONTRIBUTION: The study provides evidence for the rational use of antipsychotic medications in sub-Saharan Africa.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10912972/

Record 318

pmcid: PMC11326094

pmid: 39328862

doi: 10.12998/wjcc.v12.i27.6070

title: Application effect of case management nursing based on patient safety in patients with prostate cancer.

authors

- Zhou R
- Xu CL

journal: World journal of clinical cases

journalAbbr: World J Clin Cases

year: 2024

pubdate: 2024 Sep 26

volume: 12

issue: 27

pages: 6070-6076

abstract: BACKGROUND: Globally, prostate cancer has become a major threat to men's health, with an increasing incidence and causes serious effects on the quality and length of life of patients. Despite the rapid development of medical technology, which provides treatments, including surgery, radiotherapy, and endocrine therapy, the treatment of patients with prostate cancer, especially with endocrine therapy, has become a major challenge in clinical treatment owing to the lengthy course of treatment, side effects of drugs, and impact of the disease on the psychological and physiological functioning of the patient, producing poor treatment adherence and a decline in quality of life. AIM: To explore effects of nursing intervention prioritizing patient safety and case management in patients with prostate cancer undergoing endocrine therapy. METHODS: Eighty patients with prostate cancer who received endocrine therapy at our hospital between January 2022 and January 2024 were divided into observation and control groups with 40 cases per group. The control group was treated using a routine nursing workflow while the observation group received case management nursing guidance prioritizing patient safety. Scores for anxiety and depression, prostate cancer symptoms, and quality of life and patient compliance and satisfaction were compared between the groups after three months of intervention. RESULTS: After the nursing intervention, the anxiety and depression scores of the observation group were significantly lower than those of the control group (P < 0.05). The quality of life score, sexual function, and hormone function were significantly higher than those in the control group (P < 0.05). CONCLUSION: Case management guidance based on patient safety effectively reduced anxiety and depression in patients undergoing endocrine therapy for prostate cancer and improved their quality of life, treatment compliance, and satisfaction.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11326094/

Record 319

pmcid: PMC11182637

pmid: 38894952

doi: 10.1159/000538596

title: Arbeitsgemeinschaft Gynäkologische Onkologie Recommendations for the Diagnosis and Treatment of Patients with Early Breast Cancer: Update 2024.

authors

- Park-Simon TW
- Müller V
- Albert US
- Banys Paluchowski M
- Bauerfeind I
- Blohmer JU
- Budach W
- Dall P
- Ditsch N
- Fallenberg EM
- Fasching PA
- Fehm T
- Friedrich M
- Gerber B
- Gluz O
- Harbeck N
- Hartkopf AD
- Heil J
- Huober J
- Jackisch C
- Kolberg-Liedtke C
- Kreipe HH
- Krug D
- Kühn T
- Kümmel S
- Loibl S
- Lüftner D
- Lux MP
- Maass N
- Mundhenke C
- Reimer T
- Rhiem K
- Rody A
- Schmidt M
- Schneeweiss A

- Schütz F
- Sinn HP
- Solbach C
- Solomayer EF
- Stickeler E
- Thomssen C
- Untch M
- Witzel I
- Wuerstlein R
- Wöckel A
- Janni W
- Thill M

journal: Breast care (Basel, Switzerland)

journalAbbr: Breast Care (Basel)

year: 2024

pubdate: 2024 Jun

volume: 19

issue: 3

pages: 165-182

abstract: INTRODUCTION: Each year the interdisciplinary AGO (Arbeitsgemeinschaft Gynäkologische Onkologie, German Gynecological Oncology Group) Breast Committee on Diagnosis and Treatment of Breast Cancer provides updated state-of-the-art recommendations for early and metastatic breast cancer. METHODS: The updated evidence-based treatment recommendations for early and metastatic breast cancer have been released in March 2024. RESULTS AND CONCLUSION: This paper concisely captures the updated recommendations for early breast cancer chapter by chapter.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11182637/

Record 320

pmcid: PMC11478873

pmid: 39405302

doi: 10.1371/journal.pgph.0003736

title: Are we preparing healthy & responsible adolescents? Exploratory qualitative study to understand the health and social issues of adolescent living in Karachi, Pakistan.

authors

- Rizvi N
- Saleem S
- Mukhtar Ahmed J
- Reza SE
- Jabeen R
- Jessani S

journal: PLOS global public health

journalAbbr: PLOS Glob Public Health

year: 2024

pubdate: 2024

volume: 4

issue: 10

pages: e0003736

abstract: This study aimed to understand the lives of adolescents living in squatter settlements of Karachi, Pakistan regarding their assigned roles and responsibilities, health and social issues, their decision-making process, and mechanism and channels of information. An exploratory descriptive qualitative study design was employed. Purposive sampling techniques were used to carry out Focus Group Discussions (FGDs) with adolescents (n = 10, Participants = 190), adolescents' parents (n = 10, Participants = 180), and In-Depth Interviews of with key adolescent stakeholders (n = 20). Adolescent stakeholder mapping was conducted for enrolling participants. The data were analysed thematically using deductive and inductive approaches. Based on gender norms parents assign specific roles and responsibilities to adolescent girls and boys. Due to societal norms, communication gap exists between adolescents and their parents. The most popular information channels are social media platforms and friends. Adolescents reported being subjected to a variety of physical, sexual, mental, social, and environmental pressures. Lack of guidance from parents, inadequate knowledge and skills to deal with physical, sexual, mental, social and environmental hazards, and misuse of social media lead to risky decisions, injuries, and social instability. This study underlines the urgent need for targeted interventions for addressing gender issues and improving adolescents' decision-making and life skills. We recommend Behavior Change Communication interventions to dismantle gender stereotypes and support a balanced domestic environment for children's education and well-being, awareness raising among parents, teachers, and healthcare providers about adolescent risks emphasizing their role in youth guidance, and advocacy for youth-led forums to co-create educational content engaging parents, educators, and health professionals focusing on life skills. These strategies would turn demographic transitions into productive dividends.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11478873/

Record 321

pmcid: PMC11641981

pmid: 39685668

doi: 10.3390/jcm13237209

title: Assessing Fat Grafting in Breast Surgery: A Narrative Review of Evaluation Techniques.

authors

- Bogdan RG
- Helgiu A
- Cimpean AM
- Ichim C
- Todor SB
- Iliescu-Glaja M
- Bodea IC
- Crainiceanu ZP

journal: Journal of clinical medicine

journalAbbr: J Clin Med

year: 2024

pubdate: 2024 Nov 27

volume: 13

issue: 23

pages:

abstract: Fat grafting has gained prominence in reconstructive and aesthetic surgery, necessitating accurate assessment methods for evaluating graft volume retention. This paper reviews various techniques for assessing fat and fat grafts, including their benefits and limitations. Three-dimensional (3D) scanning offers highly accurate, non-invasive volumetric assessments with minimal interference from breathing patterns. Magnetic resonance imaging (MRI) is recognized as the gold standard, providing precise volumetric evaluations and sensitivity to complications like oil cysts and necrosis. Computed tomography (CT) is useful for fat volume assessment but may overestimate retention rates. Ultrasonography presents a reliable, non-invasive method for measuring subcutaneous fat thickness. Other methods, such as digital imaging, histological analysis, and weight estimation, contribute to fat graft quantification. The integration of these methodologies is essential for advancing fat graft assessment, promoting standardized practices, and improving patient outcomes in clinical settings.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11641981/

Record 322

pmcid: PMC11633068

pmid: 39410773

doi: 10.1177/02841851241287924

title: Assessment of pediatric breast ultrasound less is more: a practical imaging approach.

authors

- Giannotti E
- Sun R
- Healy N
- Kilburn-Toppin F
- Sofia C
- Lee AH
- Marino MA

journal: Acta radiologica (Stockholm, Sweden : 1987)

journalAbbr: Acta Radiol

year: 2024

pubdate: 2024 Dec

volume: 65

issue: 12

pages: 1465-1472

abstract: BACKGROUND: Breast cancer in pediatric patients is rare, but ultrasound (US) is widely utilized for symptomatic cases. PURPOSE: To determine biopsy and cancer detection rates of pediatric patients and to assess if breast US can be omitted. MATERIAL AND METHODS: A retrospective review of a 5-year period was conducted of single-center breast US performed in patients aged <19 years. Data regarding presentation, clinical opinion (P1-5 score), and US (U1-5 score) were collected. If biopsy or surgery was performed, pathology was reviewed (B1-5 score). RESULTS: In total, 579 patients were included (19 boys, 560 girls; mean age=16.2±1.9 years; age range=0-18 years). Clinical examination was normal or benign (P1/P2) in all boys (100%) and 557/560 (99.5%) girls, and P3 in 3 (0.5%) girls. Of US, 52% demonstrated normal findings (U1) for both sexes (300/579); in the remaining cases, the most frequent findings were gynecomastia in 12/19 boys and well-defined breast masses in 208/560 girls. Of the 560 girls, 6 (1%) underwent US-guided biopsy, with final histology of fibroadenoma (B2) in all cases, while 27 (5%) had a surgical excision, with final histology of fibroadenoma (22/27, 81.5%), hamartoma (2/27, 7.4%), benign phyllodes tumor (2/27, 7.4%), and angiomyxoma skin lesion (1/27, 3.7%). No malignant lesions were diagnosed at the time of clinical referral or during the 18-month follow-up in patients with a well-defined mass on US. CONCLUSION: Breast malignancy is extremely

rare in pediatric population. US can be safely omitted if clinical examination is normal; this approach would have avoided breast US in 52% of patients in this study.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11633068/

Record 323

pmcid: PMC11973446

pmid: 40196800

doi: 10.5001/omj.2024.100

title: Assessment of Quality of Life and Behavioral Problems in Children with Central Precocious Puberty.

authors

- Raafat S
- Abdelaal E
- Mowafi I
- Khalil M

journal: Oman medical journal

journalAbbr: Oman Med J

year: 2024

pubdate: 2024 Sep

volume: 39

issue: 5

pages: e670

abstract: OBJECTIVES: We sought to estimate the impact of central precocious puberty (CPP) on psychosocial aspects in the cohort of children and to assess whether these aspects changed after treatment with gonadotropin-releasing hormone (GnRH) analogues. METHODS: A case-control study enrolled 30 CPP children and 30 normal controls. The CPP group was assessed with emphasis on anthropometric measurements (Tanner staging for pubertal changes). The hormonal profile included gonadotropins, estradiol or testosterone, and GnRH stimulation test. The instruments used for neuropsychological assessment included the Arabic version of the pediatric quality of life inventory TM 4.0, the Child Behavior Checklist (CBCL), and IQ testing. RESULTS: The mean study group age was 5.1 ± 2.3 years compared to 5.2 ± 2.2 years in the control group; 29 patients and 29 controls (96.7%) were females. CPP children had significantly lower scores than controls in health-related quality of life (QoL) domains except school functioning without significant changes in CBCL scores and cognitive function. There was a statistically significant improvement in these scores after they were treated with GnRH analogues for a year. CONCLUSIONS: There are significant aberrations in CPP children's QoL with no effect on behavior or cognition. After treatment with GnRH analogues for a year, QoL and CBCL T-scores showed a significant improvement.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11973446/

Record 324

pmcid: PMC11532728

pmid: 39498284

doi: 10.1016/j.jpra.2024.09.011

title: Available techniques to minimize scars in surgical management of gynecomastia - a comprehensive review.

authors

- Frigerio G
- Serre A

- Engels PE
- Kalbermatten DF
- André-Lévigne D

journal: JPRAS open

journalAbbr: JPRAS Open

year: 2024

pubdate: 2024 Dec

volume: 42

issue:

pages: 256-267

abstract: OBJECTIVE: Gynecomastia and lipomastia are benign proliferations of the male breast affecting 32-65% of men. Numerous surgical procedures often result in stigmatizing scars when it comes to skin resection. The purpose of this study was to review skin-sparing techniques and to describe our skin-sparing approach to treat skin excess using transcutaneous netting. MATERIALS AND METHODS: A comprehensive review of the literature was conducted aiming at identifying available techniques to avoid skin resection in gynecomastia or lipomastia patients (Simon's grade IIb and III). Surgical techniques, patient satisfaction, time of follow-up, and complications were assessed. RESULTS: Seven studies detailed skin retraction techniques, including laser-, ultrasound-, and radiofrequency-assisted liposuction (LAL, UAL, and RAL), microneedling, and nipple-areolar complex (NAC) plaster lifting. All articles provided Simon's grade classification, with most studies including patients with and without skin laxity. Complication rates were low (1.5-10%), and patient satisfaction ranged from 87.5% to 100%.While transcutaneous netting has been reported to reduce hematoma in gynecomastia surgery, no studies specifically examined its role in managing skin redundancy. CONCLUSIONS: Limited data exist on scarless skin retraction techniques for gynecomastia. While LAL, UAL, and RAL show some potential, controlled studies are lacking, and skin resection is often performed for high skin redundancy. We recommend a skin-sparing approach using liposuction and transcutaneous netting for gynecomastia up to Simon's grade III, which allows for skin retraction and NAC fixation. No literature was found assessing the efficacy of transcutaneous netting in promoting skin retraction in gynecomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11532728/

Record 325

pmcid: PMC11537988

pmid: 39507494

doi: 10.3389/fped.2024.1417050

title: Benign and malignant breast lesions in children and adolescents - diagnostic and therapeutic approach.

authors

- Sosnowska-Sienkiewicz P
- Januszkiewicz-Lewandowska D
- Mańkowski P

journal: Frontiers in pediatrics

journalAbbr: Front Pediatr

year: 2024

pubdate: 2024

volume: 12

issue:

pages: 1417050

abstract: Benign and malignant breast lesions in children and adolescents are rare compared to adults. Most tumors are benign. Malignant breast lesions are extremely rare. Fibroadenomas are the most common, accounting for 95% of all lesions. Diagnosis is based on history and physical examination of the breast and armpit. Imaging studies include ultrasound, mammography, and magnetic resonance imaging. Ultrasound is the most commonly used imaging test. Other tests are used in cases of diagnostic doubt. Core needle biopsy should be considered for appropriate diagnostic management. Excisional biopsy should be considered for complex clinical conditions and imaging studies. Except in doubtful situations in children and adolescent girls, a conservative approach and observation of the lesions along with periodic ultrasound examination initially every 6-12 months is advisable. Management of malignant breast lesions in children typically involves a multidisciplinary team consisting of pediatric oncologists, surgeons, radiation oncologists, pathologists, and other specialists and depends on the clinical condition of the patient. An important aspect is the experience of the clinician and radiologist in the treatment of breast lesions, as well as increasing patient and family awareness of possible breast lesions and self-examination. This review aims to provide a scoping overview of the available literature on benign and malignant lesions of the breast in pediatric and adolescent populations to assist physicians and surgeons in making decisions regarding the appropriate diagnosis and management of pediatric breast disease.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11537988/

Record 326

pmcid: PMC11367847

pmid: 39218906

doi: 10.1186/s13256-024-04707-9

title: Bilateral breast metastases from anaplastic lymphoma kinase-positive lung cancer in a male: a case report.

authors

- Koh S
- Koi Y
- Tajiri W
- Kawasaki J
- Akiyoshi S
- Nakamura Y
- Koga C
- Okamoto T
- Taguchi K
- Tokunaga E

journal: Journal of medical case reports

journalAbbr: J Med Case Rep

year: 2024

pubdate: 2024 Sep 2

volume: 18

issue: 1

pages: 402

abstract: BACKGROUND: Distant metastases from lung cancer are commonly found in the brain, bone, and liver. Metastases to the breast from non-mammary malignancies are extremely rare, and their clinical presentations remain unclear. CASE PRESENTATION: We herein report a case of bilateral breast metastases from anaplastic lymphoma kinase-positive advanced lung cancer in a 51-year-old Japanese male patient. During the course of systemic treatment for advanced lung cancer, computed tomography revealed bilateral

breast enlargement without contrast enhancement, a finding consistent with gynecomastia. While other metastatic lesions responded to chemotherapy, both breast masses grew vertically like nodules. The breast masses were immunohistochemically diagnosed as metastases from lung cancer and were removed surgically. Simultaneous bilateral breast metastases from malignancies of other organs, like ones in this case, have rarely been described. CONCLUSIONS: It is important to keep in mind that breast metastases from nonmammary malignancies are a possible explanation for unusual breast findings in a patient with a history of malignancies.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11367847/

Record 327

pmcid: PMC10906603

pmid: 38435464

doi: 10.1097/GOX.0000000000005639

title: Bilateral Hematoma Following Air Travel after Breast Augmentation: A Cautionary Tale and Literature Review.

authors

- Kyriazidis I
- Antoniou A
- Demiri E
- Pavlidis L
- Papas A

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2024

pubdate: 2024 Mar

volume: 12

issue: 3

pages: e5639

abstract: In this study, we report a rare incidence of bilateral hematoma following air travel in a patient who had recently undergone breast augmentation surgery. The case underscores the potential risks associated with flying shortly after such procedures. Through a literature review, we aimed to explore the incidence rates and highlight the need for further research in this area. This case report aims to raise awareness among healthcare professionals and patients about the need for cautious post-surgical travel planning to mitigate the risk of similar complications.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10906603/

Record 328

pmcid: PMC11787721

pmid: 39896114

doi: 10.5001/omj.2024.24

title: Bilateral Reduction Mammoplasty with Nipple-areola Complex Graft in Ten-Year-Old Girl with Juvenile Gigantomastia: A Case Report.

authors

- Ali K
- Husain M

- Alrayes A
- Alsaffar H

journal: Oman medical journal

journalAbbr: Oman Med J

year: 2024

pubdate: 2024 Jul

volume: 39

issue: 4

pages: e660

abstract: Gigantomastia is a rare benign disorder characterized by excessive breast enlargement. Although a standard definition has yet to be established, it is generally known as an excess of approximately 1.5 kg tissue per breast that requires reduction. We report a case of a 10-year-old girl who presented with enlarged breasts. The hormonal assay was normal. Breast ultrasound revealed increased fibroglandular breast tissue with an area of interstitial fluid, prominent vascularity, and dilated veins. There was no evidence of focal masses or collections, ductal ectasia, or axillary lymphadenopathy. Magnetic resonance imaging revealed extremely dense breasts with more than 75% fibroglandular tissue, consistent with juvenile breast hypertrophy. Bilateral reduction mammoplasty with nipple-areola complex graft was performed. The histopathological report confirmed benign gigantomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11787721/

Record 329

pmcid: PMC11342000

pmid: 39184929

doi: 10.21037/tbcr-24-26

title: Breast cancer in pregnancy: a comprehensive review of diagnosis, management, and outcomes.

authors

- Mumtaz A
- Otey N
- Afridi B
- Khout H

journal: Translational breast cancer research : a journal focusing on translational research in breast cancer

journalAbbr: Transl Breast Cancer Res

year: 2024

pubdate: 2024

volume: 5

issue:

pages: 21

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11342000/

Record 330

pmcid: PMC10903845

pmid: 38421972

doi: 10.1371/journal.pone.0299642

title: Breast hypoplasia markers among women who report insufficient milk production: A retrospective online survey.

authors

- Kam RL
- Amir LH
- Cullinane M
- Ingram J
- Li X
- Nommsen-Rivers LA

journal: PloS one

journalAbbr: PLoS One

year: 2024

pubdate: 2024

volume: 19

issue: 2

pages: e0299642

abstract: OBJECTIVES: To estimate the proportions of anatomical breast characteristics suggestive of breast hypoplasia among breastfeeding women self-reporting low milk supply. We also explored breast hypoplasia risk factors. DESIGN: Online survey conducted between October 2021 and January 2022. SETTING: Five low milk supply Facebook groups. PARTICIPANTS: 487 women reporting low milk supply with their first child born ≥ 37 weeks gestation within 5 years of participation in this study, and residing in the USA, Australia or the UK. We present data on the primary outcome ('breast type') for 399 women. Women were excluded if the dyad was separated for more than 24 hours during the hospital stay, or if the mother reported removing milk less than 6 times per day from each breast on most days before being aware of having insufficient milk production. PRIMARY AND SECONDARY OUTCOME MEASURES: The proportions of proposed breast hypoplasia markers including atypical breast type, widely spaced breasts, breast asymmetry, stretch marks on the breast and lack of pregnancy breast growth. We also estimated the odds of having breast hypoplasia markers in at-risk groups compared to reference groups, adjusting for covariates. RESULTS: Approximately 68% reported at least one atypical breast (270/399; 95% CI: 62.9%, 72.1%). Around 47% reported widely spaced breasts (212/449; 95% CI: 42.7%, 52.7%), 72% a lack of pregnancy breast growth (322/449; 95% CI: 68.3%, 77.4%), and 76% stretch marks on the breast (191/250; 95% CI: 70.7%, 81.3%). Multiple logistic regression analyses identified being overweight during pubertal years as a risk factor for atypical breast type and lack of pregnancy breast growth. CONCLUSIONS: Participants in low milk supply Facebook groups reported high rates of breast hypoplasia markers. Being overweight during adolescence was a risk factor for breast hypoplasia markers. These findings should be confirmed in well-conducted large cohort studies to determine the strongest combination of hypoplasia markers in predicting low supply.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10903845/

Record 331

pmcid: PMC11595676

pmid: 39598292

doi: 10.3390/life14111494

title: Breast Implant-Associated Anaplastic Large Cell Lymphoma: A Case Report About a Patient with Cytology Negative for Malignancy.

authors

- Ün D
- Rohrbeck J
- Drach M
- Ullrich R
- Staber PB
- Helbich TH
- Freystätter C
- Teufelsbauer M
- Radtke C

journal: Life (Basel, Switzerland)

journalAbbr: Life (Basel)

year: 2024

pubdate: 2024 Nov 16

volume: 14

issue: 11

pages:

abstract: Breast implant-associated anaplastic large cell lymphoma (BIA-ALCL) is a rare lymphoma primarily linked to textured breast implants. Symptoms are often non-specific (e.g., breast swelling, pain, or fluid collection). When imaging detects fluid around the implant, cytological examination is the first diagnostic approach. However, this method has limited sensitivity and may yield false-negative results. In this case, a 41-year-old woman presented with swelling, pain, and itching in her left breast six years after bilateral textured breast implant placement. Ultrasonography (US) revealed peri-implant fluid collection around the left implant. A following magnetic resonance imaging (MRI) scan ruled out an implant rupture. Due to persistent pain and the peri-implant effusion on the left side, open surgery was performed. During implant removal, the seroma was drained, and multiple suspicious masses were found on the left side. The cytology of the seroma fluid was negative and intraoperative frozen sections of the excised masses were inconclusive. A complete capsulectomy was conducted due to the suspicion of malignancy. Histological examination ultimately confirmed the diagnosis of BIA-ALCL. This case highlights the diagnostic challenges associated with this rare condition. Therefore, BIA-ALCL should always be considered in the differential diagnosis of breast implant-associated seroma.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11595676/

Record 332

pmcid: PMC11577858

pmid: 39563285

doi: 10.1186/s12891-024-07978-x

title: Breast size, thoracic kyphosis, and thoracic spine pain: a correlational survey of Nigerian postpartum mothers.

authors

- Petronilla OC
- Chinecherem EP
- Sunday ES

journal: BMC musculoskeletal disorders

journalAbbr: BMC Musculoskelet Disord

year: 2024

pubdate: 2024 Nov 20

volume: 25

issue: 1

pages: 934

abstract: BACKGROUND: Thoracic spine postural dysfunctions are common postpartum-related health problems, compromising breastfeeding efficacy and quality of life among women. Previous studies have particularly associated these conditions with increased breast sizes in several populations. However, such empirical evidence is scarce in the Nigerian population. OBJECTIVES: To investigate the relationship among breast size, thoracic-kyphosis, and -spine pain among postpartum Nigerian women. METHODS: This correlational survey involved 400 consenting postpartum mothers (between 0 and 24 months of postpartum period). Their breast size, thoracic spine posture, and pain were measured using a measuring tape (cm), inclinometer, and Revised Oswestry thoracic spine pain disability questionnaire, respectively. Data were analyzed using descriptive and relevant inferential statistics at $p < 0.05$. RESULTS: The majority of the participants fall under the category of breast cup size B (61.75%), have no history of thoracic spine pain (87.4%), and about half of them (50.2%) have normal thoracic spine posture (low category with values ranging between 20^0 and 35^0. Breast size was significantly ($r = 0.162$, $p = 0.001$) correlated with thoracic spine posture but showed no significant correlation ($r = 0.066$, $p = 0.622$) with thoracic spine pain. CONCLUSION: Increasing breast size is weakly associated with a tendency towards a kyphotic posture of the thoracic spine. Postural education and care around adequate support of the breast with suitable fitting brassieres may help prevent kyphotic deformities. Future research with a randomized control trial and long-term follow-up is recommended to further confirm the causal relationship of these variables.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11577858/

Record 333

pmcid: PMC11558302

pmid: 39544974

doi: 10.21037/gs-24-185

title: Breast tuberculosis with bone destruction mimicking breast cancer with bone metastasis: a case report and literature review.

authors

- Li LX
- Wang YW
- Lin Y
- Chen YD
- Chen X
- Li X
- Zhou XL
- Su L
- Lin MY
- Zhang K

journal: Gland surgery

journalAbbr: Gland Surg

year: 2024

pubdate: 2024 Oct 31

volume: 13

issue: 10

pages: 1853-1862

abstract: BACKGROUND: Tuberculosis (TB) poses a significant global health challenge. While the incidence of breast TB (BTB) is relatively low, it can easily be mistaken for breast cancer or breast granulomatous lobulitis, potentially delaying timely intervention. The gold standard for diagnosis consists of Mycobacterium TB culture and Ziehl-Neelsen staining. Treatment typically consists of a combination of anti-TB therapy and surgical intervention. CASE DESCRIPTION: Our research group recently reported a case of BTB accompanied by rib bone destruction. Initially, imaging examinations suggested breast cancer with bone metastasis. However, subsequent real-time ultrasound-guided biopsy revealed chronic inflammation with granulomatous inflammation and extensive necrosis, which led to the diagnosis of BTB. Further T cells spot test (T-SPOT) testing predicted the likelihood of the presence of TB infection, and magnetic resonance imaging (MRI) indicated a high likelihood of TB. A biopsy of the breast lesions was performed and subsequent polymerase chain reaction (PCR) analysis of the aspirated specimens confirmed the presence of DNA from the Mycobacterium TB complex, as well as genes associated with rifampicin resistance. These findings led to the definitive diagnosis of BTB complicated by bone destruction. The patient underwent a 4-month course of anti-TB medication, followed by surgical intervention and an additional regimen of anti-TB drugs. Nine months post-surgery, the patient exhibited no indications of recurrence. CONCLUSIONS: The diagnosis of BTB with bone destruction is challenging, and clinicians need to be vigilant not to misdiagnose it as breast cancer or granulomatous lobulitis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11558302/

Record 334

pmcid: PMC11059175

pmid: 39555926

doi: 10.5152/FNJN.2024.23231

title: Breastfeeding After Breast Augmentation Surgery: A Scoping Review.

authors

- Kazankaya F
- Paker SA
- Demirel Bozkurt Ö

journal: Florence Nightingale journal of nursing

journalAbbr: Florence Nightingale J Nurs

year: 2024

pubdate: 2024 Feb 28

volume: 32

issue: 1

pages: 99-109

abstract: The aim of this scoping review was to provide a comprehensive overview of research in the literature on breastfeeding experiences of mothers undergoing breast augmentation surgery and the factors influencing this, and to identify gaps in the literature that could inform future design and evaluation. The study was conducted as a comprehensive review based on the approach of Arksey and O'Malley. After scans of relevant databases, such as Scholar, Information Sciences Institute, Science Direct, Ovid, PubMed, and Scientific Information Database, all data were collected, summarized, and given a narrative explanation. In the literature, the correlation between topics involving women's health and aesthetic breast surgery could not be clearly shown. When certain illnesses appeared, aesthetic surgery was blamed, and women who had undergone breast augmentation surgery were undecided about breastfeeding when they became mothers. It was determined in this study that the effects on breastfeeding success in women who have undergone breast augmentation surgery fall principally under three headings: surgery type, placement of implant material, and surgery complications. It is crucial to provide detailed information about the possible consequences of this surgery to

women of reproductive age considering this surgery.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11059175/

Record 335

pmcid: PMC10726215

pmid: 38111716

doi: 10.1016/j.jpra.2023.11.007

title: Capsular pneumatosis: A rare radiographic sign for internal breast implant capsule violation in trauma.

authors

- Lemoine C
- Ramos HD
- Healy A
- Babigian A

journal: JPRAS open

journalAbbr: JPRAS Open

year: 2024

pubdate: 2024 Mar

volume: 39

issue:

pages: 75-80

abstract: Breast augmentation is considered one of the most commonly performed procedures by aesthetic plastic surgeons, representing 16 % of all global plastic surgery procedures in 2020. Given the fact that thoracic trauma comprises over 20 % of trauma worldwide, it is unsurprising that there is potential for overlap between these two patient populations. Here, we present the case of a 59-year-old patient who had undergone bilateral breast augmentation over 10 years prior to presentation. They arrived as a highest-level trauma activation after being a helmeted cyclist struck by a motor vehicle resulting in significant left-sided thoracic trauma. Following stabilization in the trauma bay, CT imaging of the thorax demonstrated multifocal left pulmonary contusions and lacerations, multiple left-sided rib fractures (ribs 2-12), a small left pneumothorax, and left-sided subcutaneous emphysema. Imaging also demonstrated the presence of bilateral breast implants with the left implant appearing irregular in shape with the retropectoral space corresponding to the implant capsule having evidence of significant free air (capsular pneumatosis) concerning for acute traumatic rupture of the capsule. While undergoing surgical stabilization of her left-sided rib fractures, one of her ribs was noted to have violated the posterior wall of the breast capsule. Upon implant removal, the implant was found to have ruptured with tears in the shell corresponding to patient's rib fractures. This case represents a rare and unexpected complication of traumatic rib fractures; mainly the traumatic rupture of a silicone breast implant, which was identified by the presence of capsular pneumatosis on CT imaging. Presence of this rare radiographic sign (capsular pneumatosis) in the setting of a patient who has undergone breast augmentation should raise concern for possible implant rupture and capsule violation, even in the absence of external signs of penetrating injury.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10726215/

Record 336

pmcid: PMC10839428

pmid: 38318566

doi: 10.7759/cureus.51641

title: Centrifugally Spreading Annular Erythema as a Dermatological Indicator of Metastatic Breast Carcinoma.

authors

- Youh J
- Yamaguchi Y
- Hiraguchi E

journal: Cureus

journalAbbr: Cureus

year: 2024

pubdate: 2024 Jan

volume: 16

issue: 1

pages: e51641

abstract: Breast cancer is the leading cause of skin metastasis in women with internal malignancies. This report highlights an atypical case of cutaneous metastasis of breast cancer (CMBC) in a 66-year-old woman. Starting four months before her dermatology consultation, the patient underwent a chemotherapy regimen comprising pertuzumab, trastuzumab, and vinorelbine for right breast cancer, right axillary lymph node enlargement, and bone metastases. After commencing chemotherapy, erythematous macules appeared around her right nipple. Subsequently, the cutaneous lesions developed into annular erythematous patches around her right nipple and began to coalesce and expand to the contralateral breast. A skin biopsy revealed dysplastic cells indicative of metastasis from invasive ductal carcinoma. In addition, lymphovascular tumor cell invasion was noted in the reticular dermis. Based on these clinical progressions and histopathologic findings, a diagnosis of CMBC was made, specifically considering the possibility of inflammatory breast cancer (IBC). The patient continued the same chemotherapy regimen for 17 cycles, which improved the skin lesions, but she succumbed to breast cancer two years later. This case emphasizes the importance of considering CMBC in breast cancer patients with expanding, treatment-resistant thoracic cutaneous lesions, especially in aggressive subtypes like IBC. The diverse presentations of CMBC require thorough histopathological evaluation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10839428/

Record 337

pmcid: PMC11045276

pmid: 38665331

doi: 10.1155/2024/9987144

title: Challenges in Management of Ovotesticular Differences in Sex Development in Resource-Limited Settings.

authors

- Asafo-Agyei SB
- Ameyaw E
- Nimako B
- Amoah M

journal: Case reports in endocrinology

journalAbbr: Case Rep Endocrinol

year: 2024

pubdate: 2024

volume: 2024

issue:

pages: 9987144

abstract: Differences in sex development (DSD) are congenital conditions in which the development of chromosomal, gonadal, or anatomical sex is atypical. Ovotesticular DSD is the rarest variant of DSD where both ovarian and testicular tissues co-exist in an individual. Ambiguous genitalia may be a glaring indicator of DSD, but multiple genital anomalies should also raise a suspicion of DSD. This is a case report of a 15-year-6-month-old boy who presented during infancy with multiple genital anomalies requiring surgery. The diagnosis of ovotesticular DSD was missed until later in adolescence when he presented with progressive bilateral breast enlargement. Work-up revealed a 46, XX karyotype and dysgenetic testes, but functional ovarian tissue. The patient wanted to consider switching to a female gender but was constrained by psychosocial factors. Maintenance of a masculine phenotype was done using testosterone injections due to the relatively high cost of testosterone patches. Conclusion . Multiple genital anomalies should raise the suspicion of DSD, and prompt referral to an endocrinologist should be done before urogenital surgery and gender assignment are carried out.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11045276/

Record 338

pmcid: PMC11542342

pmid: 39512981

doi: 10.7759/cureus.71071

title: Clinical Insights Into Ductal Carcinoma In Situ in Males: A Report of Two Cases From the Sultanate of Oman.

authors

- Eshaqi MJ
- Al Shamsi SA
- Al Saidi AM
- Mahesh S

journal: Cureus

journalAbbr: Cureus

year: 2024

pubdate: 2024 Oct

volume: 16

issue: 10

pages: e71071

abstract: Breast cancer is a disease that predominantly affects the female population; however, rarely it can manifest in males, yet its etiology remains poorly elucidated. The scarcity of literature reviews and case reports done on male breast cancer in comparison to the female counterpart makes it difficult to understand the risk factors, treatment options, and extension of the disease. Moreover, high-grade ductal carcinoma in situ (DCIS) is exceptionally uncommon among male patients. The prognosis for male patients diagnosed with DCIS is similar to that of females at the same disease stage making early recognition and diagnosis significant. However, more efforts are being made to understand the clinical presentation, increase awareness, and acknowledge the etiology of this disease. This study addresses this gap by presenting two distinctive cases of invasive ductal carcinoma in males from the Sultanate of Oman. The aim of this study is to contribute to the growing efforts in comprehending the unclear landscape of male breast cancer, fostering awareness, and advancing knowledge of its etiology.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11542342/

Record 339

pmcid: PMC11500512

pmid: 39465086

doi: 10.1080/26895269.2023.2284775

title: Clinical management of transgender and non-binary patients in the fertility preservation service: Current evidence.

authors

- Tsonis O
- Kopeika J

journal: International journal of transgender health

journalAbbr: Int J Transgend Health

year: 2024

pubdate: 2024

volume: 25

issue: 4

pages: 663-680

abstract: Background: Transgender and non-binary individuals face unique challenges when it comes to fertility preservation (FP). Objective: Despite the growing prevalence of gender dysphoria (GD) and gender transitioning, there is a lack of clear guidelines and consensus on the management of these patients in the FP setting. Clinicians and institutions providing FP services should ensure that they are aware of the needs and circumstances of this underrepresented group of patients and offer them accurate and evidence-based information when counseling and tailoring their FP treatment. Materials and methods: For this scoping review, three major search engines were used. Including Embase, Epistemonikos, Google Scholar, MEDLINE and PubMed. Sources of grey literature were also explored (ResearchGate and Web of Science). The combination of only two keywords [transgender] AND [fertility preservation] was used up to May 2023. Results: The available evidence on clinical management and FP outcomes in transgender patients is limited and mainly originates from case reports or small case series. The main limitation of current FP services for transgender and non-binary individuals is the lack of scientific evidence regarding their care. Discussion: Overall, FP in transgender patients requires individualized and realistic plans, and psychological counseling should be offered. This review aims to provide the latest evidence coming from original studies to facilitate proper counseling and fertility management for these individuals. Conclusions: Inclusive health systems that provide comprehensive reproductive health care to transgender individuals can help them make informed decisions about FP and improve their quality of life. Future research is needed to establish more robust evidence-based guidelines for the management of transgender and non-binary individuals in the FP setting.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11500512/

Record 340

pmcid: PMC11602333

pmid: 39610685

doi: 10.3389/fmed.2024.1442525

title: Commonly encountered symptoms and their management in patients with cirrhosis.

authors

- Philips CA

journal: Frontiers in medicine

journalAbbr: Front Med (Lausanne)

year: 2024

pubdate: 2024

volume: 11

issue:

pages: 1442525

abstract: This exhaustive review, explored the multifaceted symptoms and their management in patients with cirrhosis. Patients frequently endure pain, muscle cramps, sleep disturbances, psychological distress, and gastrointestinal issues, significantly impairing their quality of life. Pain is prevalent, often requiring analgesics, while muscle cramps affect up to 68% of patients, treated with supplements like zinc and taurine despite limited evidence. Sleep disturbances, including insomnia and excessive daytime sleepiness, afflict up to 80% of patients, managed through lactulose, melatonin, and cognitive behavioral therapies. Gastrointestinal symptoms, affecting 80%, include abdominal pain and bloating, necessitating lifestyle and dietary adjustments. Mental health disorders, such as depression and anxiety, are common, managed with a combination of pharmacotherapy and psychotherapy. Sexual dysfunction, often overlooked, profoundly impacts both men and women, requiring holistic treatment approaches. Pruritus, another distressing symptom, is managed with moisturizers and antihistamines, though many treatments show limited success. Hair loss and skin changes add to the psychological burden, highlighting the need for a comprehensive, multidisciplinary approach. The review underscores the imperative for tailored, compassionate care to enhance patient outcomes and quality of life in cirrhosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11602333/

Record 341

pmcid: PMC10847174

pmid: 38147295

doi: 10.1007/s42000-023-00510-1

title: Comparison of clinical characteristics of a pediatric cohort with combined pituitary hormone deficiency caused by mutation of the PROP1 gene or of other origins.

authors

- Zygmunt-Górska A
- Wójcik M
- Gilis-Januszewska A
- Starmach A
- Bik-Multanowski M
- Starzyk JB

journal: Hormones (Athens, Greece)

journalAbbr: Hormones (Athens)

year: 2024

pubdate: 2024 Mar

volume: 23

issue: 1

pages: 69-79

abstract: UNLABELLED: The most commonly identified genetic cause of combined pituitary hormone deficiency (CPHD) is PROP1 gene mutations. The aim of the study was to compare selected clinical features of patients with CPHD caused by variants of the PROP1 gene (CPHD-PROP1) and patients with inborn CPHD of other etiology (CPHD-nonPROP1). MATERIAL AND METHODS: The retrospective analysis included childhood medical records of 74 patients (32 female) with CPHD, including 43 patients (23 female) with the mutation in the PROP1 gene. RESULTS: Patients with CPHD-PROP1 compared to the CPHD-nonPROP1 presented with the following: significantly higher median birth weight (0.21 vs. - 0.29 SDS, p = 0.019), lower growth velocity within 3 years preceding growth hormone administration (- 2.7 vs. - 0.8 SDS, p < 0.001), higher mean maximal blood concentration of growth hormone within the stimulation process (1.2 vs. 1.08 ng/mL, p = 0.003), lower TSH (1.8

vs. 2.4 µIU/mL, p < 0.001), significantly lower prolactin concentrations (128 vs. 416.3 µIU/mL, p < 0.001), and less frequent typical signs of hypogonadism at birth in boys (n = 6; 30% vs. n = 12, 54%, p < 0.001). Secondary adrenal insufficiency was less frequent in CPHD-PROP1 (20 vs. 25 cases, p = 0.006) and occurred at a later age (13.4 vs. 10.4 years). MRI of the pituitary gland in CPHD-PROP1 revealed a small pituitary gland (21 cases), pituitary gland enlargement (eight cases), and one pituitary stalk interruption and posterior lobe ectopy, while it was normal in nine cases. CONCLUSION: Patients with the PROP1 mutations present a clinical picture significantly different from that of other forms of congenital hypopituitarism. Certain specific clinical results may lead to the successful identification of children requiring diagnostics for the PROP1 gene mutation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10847174/

Record 342

pmcid: PMC10879692

pmid: 38389650

doi: 10.1016/j.jpra.2024.01.009

title: Complications after breast augmentation with dermal fillers containing copolyamide: A systematic review.

authors

- Hedström K
- Falk-Delgado A
- Sackey H

journal: JPRAS open

journalAbbr: JPRAS Open

year: 2024

pubdate: 2024 Jun

volume: 40

issue:

pages: 19-31

abstract: BACKGROUND: Dermal fillers containing copolyamide are used for breast augmentation and are marketed under different labels, such as Aquafilling, Los Deline, Aqualift, and Activegel. In recent years, the number of publications reporting complications after use of these fillers has increased. METHODS: Through a computerized search following the Preferred Reporting Items for Systematic Reviews and Meta-Analyses guidelines, a systematic review of published studies on complications, treatment options, and radiological findings related to breast augmentation with dermal fillers containing copolyamide was performed. Publications between January 1, 2007, and January 23, 2023, were included. Retrieved studies were screened for inclusion and quality assessment. The Joanna Briggs checklist for case reports and the Strengthening the Reporting of Observational Studies in Epidemiology checklist for cross-sectional studies were used. RESULTS: Sixteen studies met the inclusion criteria: 14 case reports and 2 retrospective cohort studies, including 196 women and 333 complications. Long-term complications (≥30 days after surgery) were described in 15 studies. The most commonly reported complications were nodules in the breast (130 patients), pain (92 patients), inflammation and/or infection (43 patients), breast deformities (35 patients), and migration of the filler to the pectoralis muscle, abdominal wall, thoracic wall, pubic area, back, or upper extremity (27 patients). The median time between injection of the dermal filler and any complication was 18 months, and the majority of patients with complications required surgical intervention. CONCLUSION: Given the reports of severe complications months to years after injection of dermal fillers containing copolyamide and the lack of studies evaluating long-term safety, our interpretation is that dermal fillers containing copolyamide should not be used for breast augmentation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10879692/

Record 343

pmcid: PMC10844178

pmid: 38327867

doi: 10.1007/s13197-023-05722-2

title: Conventional and Scientific uses of Rice-washed water: A Systematic Review.

authors

- Chithambharan A
- Pottail L
- Sharma SC
- Mirle RM
- Rajalakshmi R
- Ponnusamy A

journal: Journal of food science and technology

journalAbbr: J Food Sci Technol

year: 2024

pubdate: 2024 Mar

volume: 61

issue: 3

pages: 414-428

abstract: UNLABELLED: Rice is considered the king of cereals. It is the only cereal that is being consumed by half of the population in the world. Rice and rice products have potential health benefits. One such rice aided product is rice washed water which is discussed in this article. Rice-washed water, which is commonly regarded as waste water and discarded, is a rich source of minerals and nutrients. The processing of rice washed water, nutritional analysis; edible fungi productions are detailed in this review. The article goes into detail about rice-washed water, which has been used for various purposes since our ancestors' time. The article provides a comprehensive report on the uses of rice-washed water in plant growth, Ayurveda, food, Cosmetics and a variety of other applications. Rice washed water is being utilized for various ailments since primitive times. The detailed report on the treatment utilizing rice washed water is also provided in this article. There are scarce researches with rice washed water, this article address the conventional uses of rice washed water, which can be taken up by the research community which needs more scientific validation. This review article also includes details about the composition and a variety of other important information about rice-washed water. SUPPLEMENTARY INFORMATION: The online version contains supplementary material available at 10.1007/s13197-023-05722-2.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10844178/

Record 344

pmcid: PMC10912442

pmid: 38049686

doi: 10.1007/s00266-023-03711-6

title: Cosmetic Breast Augmentation with Autologous Ex Vivo-Expanded Adipose-Derived Mesenchymal Stem/Stromal Cell (Stemform®)-Enriched Fat Grafts: A Study of the First Twenty-Two Real-World Patients.

authors

- Mamsen FPW
- Fischer-Nielsen A
- Svalgaard JD
- Jensen JD

- Jønsson B
- Duscher D
- Christensen J
- Van Leeuwen M
- Kiilerich CH
- Roider L
- Sterodimas A
- Munthe-Fog L
- Kølle ST

journal: Aesthetic plastic surgery

journalAbbr: Aesthetic Plast Surg

year: 2024

pubdate: 2024 Jan

volume: 48

issue: 2

pages: 98-115

abstract: BACKGROUND: Fat grafting is commonly utilized in breast surgery, and since it was first described, clinicians and researchers have stridden towards improvement of graft retention. Current advancements include adding adipose-derived mesenchymal stem/stromal cells (MSC(AT)s), which have demonstrated promise for improved graft retention. OBJECTIVES: This study reports outcomes for the first twenty-two patients undergoing breast augmentation (Stemform BA) or artificial implant replacement (Stemform AIR) with MSC(AT)-enriched fat in a real-world setting. METHODS: Autologous MSC(AT)s were isolated and expanded ex vivo, then mixed with lipoaspirate and injected as enriched fat for Stemform BA and AIR. The breast volume was measured preoperatively and at 3 and 12 months postoperative using a 3D Infinity Dual-Lens Camera and LifeVizApp software. Additionally, independent plastic surgeons evaluated clinical images, and patient satisfaction was obtained at equal time points. RESULTS: Twenty-two patients were included. All completed 3 and 12 months clinical follow-up and 3 months volume measurements. Nineteen patients completed 12 months volume measurements. The median fat graft retention at 12 months was 95.7% (IQR = 82.44-103.12%) for Stemform BA patients and 113.0% (IQR = 94.8-131.2%) for Stemform AIR patients. The Stemform BA patients had a median breast enlargement of 172.0% (IQR = 156.7-241.0%). The implant replacement volume of Stemform AIR patients was 102% (IQR = 85.1-130.3%). The patient reported 92.8% and 100% would elect to repeat treatment if they had the opportunity for Stemform BA and Stemform AIR, respectively. CONCLUSION: Breast augmentation and breast implant replacement patients receiving ex vivo-expanded MSC(AT)-enriched fat grafts had high graft retention and patient satisfaction scores. The paper confirms the clinical efficacy of using ex vivo-expanded MSC(AT)s. Level of Evidence V This journal requires that authors assign a level of evidence to each article. For a full description of these Evidence-Based Medicine ratings, please refer to the Table of Contents or the online Instructions to Authors www.springer.com/00266 .

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10912442/

Record 345

pmcid: PMC11157406

pmid: 38855582

doi: 10.21037/tau-24-12

title: Could menopause drug fezolinetant show promise for vasomotor symptoms associated with androgen deprivation therapy?

authors

- Qin KR

- Mertens E
- Tempo JA
- Perera M
- Brennan J

journal: Translational andrology and urology

journalAbbr: Transl Androl Urol

year: 2024

pubdate: 2024 May 31

volume: 13

issue: 5

pages: 920-922

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11157406/

Record 346

pmcid: PMC11382843

pmid: 38918655

doi: 10.31557/APJCP.2024.25.6.1945

title: Cultural Adaptation and Validation of the Punjabi Version of EPIC.

authors

- Lee TK
- Kwan W
- Singh-Carlson S

journal: Asian Pacific journal of cancer prevention : APJCP

journalAbbr: Asian Pac J Cancer Prev

year: 2024

pubdate: 2024 Jun 1

volume: 25

issue: 6

pages: 1945-1951

abstract: BACKGROUND: With earlier prostate cancer (PCa) diagnosis and increased survivorship, post-treatment quality of life (QoL) has become increasingly important. The Expanded Prostate Cancer Index Composite (EPIC) is a widely adopted QoL instrument for PCa. We aimed to create a Punjabi version of EPIC to further research in the Punjabi-speaking population. METHODS: A prototype of the Punjabi version of EPIC was created by forward-backward translations and revision. After concluding the cultural adaptation phase by interviewing 15 participants, a pilot version was created. Validation of the pilot version was performed by having 72 participants complete the Punjabi EPIC and another commonly used QoL instrument, the EORTC QLQ-c30, twice within a 4-week period. Test retest reliability (Pearson's correlations and difference distribution) and internal consistency (Cronbach's alpha) were measured using SAS version 9.4. RESULTS: Modifications were needed for the prototype Punjabi version after forward-backward translations. Cultural adaptation has highlighted a few issues including syntax and terminology. Test-retest reliability of the Urinary, Bowel, Sexual and Hormone domains were 0.88, 0.91, 0.91, and 0.95, respectively, and subscale correlations ranged from 0.75 to 0.93. Internal consistency for domains and subscales was good except for Sexual Domain. Performance of EPIC is comparable, and in some cases, slightly better than validated Punjabi version of EORTC QLQ-C30.

CONCLUSIONS: The EPIC questionnaire was successfully translated into Punjabi and was culturally adapted. The resultant Punjabi version has high reliability and validity and will be an important tool for QoL research in the Punjabi population. EPIC was successfully translated, culturally adapted, and validated with high reliability and validity into Punjabi. It will be a valuable QoL tool for physicians in clinical and research settings, and for patients in decision-making.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11382843/

Record 347

pmcid: PMC10902493

pmid: 38433789

doi: 10.1177/22925503221088847

title: Cyclic High Negative-Pressure External Volume Expansion Reduces Daily Device Application Time With Similar Effects on Recipient Site Preparation in a Murine Model.

authors

- Jeong J
- Park JK
- Bin Choy Y
- Shim JH
- Kang SM
- Nam SY
- Najmiddinov B
- Heo CY

journal: Plastic surgery (Oakville, Ont.)

journalAbbr: Plast Surg (Oakv)

year: 2024

pubdate: 2024 Feb

volume: 32

issue: 1

pages: 100-106

abstract: Introduction: Recipient site preparation using external volume expansion (EVE) increases graft survival in large-volume fat grafting. To improve patient compliance with using the device, we tested a new cyclic high negative-pressure (CHNP) mode that involves 1 h/day at -55 mm Hg, cycled between 1-second negative-pressure activation, followed by a 2-second deactivation period in an animal model. Material and Method: A miniaturized EVE device was applied to 30 8-week-old male Sprague-Dawley rats. The rats were assigned to 3 groups (no pressure for the control group, conventional -25 mm Hg for 8 h/day for conventional EVE, and CHNP mode for the CHNP group). After 28 days, micro-computed tomography was performed and skin biopsy specimens were obtained. Results: The CHNP group showed a 6.6-fold increase and the conventional EVE group showed a 4.4-fold increase in volume compared to the control group. Hematoxylin and eosin staining showed a similar increase in subcutaneous tissue thickness in both EVE groups, compared to the control group. Masson's trichome and proliferating cell nuclear antigen staining showed significantly higher collagen deposition and subdermal adipocytes in EVE groups. Immunohistochemistry against platelet endothelial cell adhesion molecule 1 showed 2.5- and 2.7-times higher vessel density in the conventional and CHNP EVE groups, respectively. There was no statistically significant difference in subcutaneous tissue thickness, collagen deposition, subdermal adipocyte proliferation, and vessel density between the 2 EVE groups. Conclusion: CHNP produced comparable results in recipient site preparation (subcutaneous tissue thickening and angiogenesis) compared to the conventional protocol, while markedly reducing the daily wear-time from 8 hours to 1 hour. Although further clinical data must be acquired, our new pressure setting seems promising and

provides a more patient-friendly pre-expansion environment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10902493/

Record 348

pmcid: PMC11745835

pmid: 39840195

doi: 10.7759/cureus.76121

title: Dasatinib-Induced Gynecomastia: A Case Report.

authors

- Kaushik H
- Devde KD
- Manuja N
- Arora R
- Dongre A

journal: Cureus

journalAbbr: Cureus

year: 2024

pubdate: 2024 Dec

volume: 16

issue: 12

pages: e76121

abstract: Gynecomastia, the abnormal enlargement of male breast tissue, is a rare side effect associated with dasatinib. This drug is used in the treatment of chronic myeloid leukemia (CML). We present a case of dasatinib-induced gynecomastia in a 52-year-old gentleman with CML who developed bilateral breast enlargement and tenderness after approximately four months of dasatinib treatment. The patient's hormonal profile was within normal limits, including testosterone, estradiol, prolactin, and follicle-stimulating hormone levels. While the exact pathophysiology remains unclear, it is postulated that dasatinib's inhibition of various kinases, including src family kinases and receptor kinases, may contribute to developing gynecomastia. The reported incidence of dasatinib-induced gynecomastia is low, and the onset of symptoms can vary widely. Management strategies for dasatinib-induced gynecomastia are not well-established, but options include androgen support, tamoxifen, or switching to an alternative tyrosine kinase inhibitor. This case report highlights the importance of monitoring patients on dasatinib therapy for developing gynecomastia and other hormonal abnormalities. Clinicians should educate the patients about the possibility of this potential adverse effect, emphasizing the need to report any symptoms indicative of low testosterone syndromes. Further research is warranted to better understand the underlying mechanisms, risk factors, and optimal management strategies for dasatinib-induced gynecomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11745835/

Record 349

pmcid: PMC11293775

pmid: 39100380

doi: 10.4103/indianjpsychiatry.indianjpsychiatry_699_23

title: Delusion of pregnancy: Case series.

authors

- Sharma M

- Singh R
- Sahu S
- Pruthi S

journal: Indian journal of psychiatry

journalAbbr: Indian J Psychiatry

year: 2024

pubdate: 2024 Jun

volume: 66

issue: 6

pages: 576-580

abstract: Delusion of pregnancy is a relatively uncommon manifestation in psychotic patients. Differentiating it from closely related clinical entities like pseudocyesis/pseudopregnancy can be a challenge in clinical setting. Diagnosis of a psychotic illness with delusional pregnancy as the presenting feature in the absence of other florid symptoms of psychosis is a different challenge altogether. Currently, there is no consensus on the pharmacotherapy and psychotherapy of delusional pregnancy. We present a case series on delusional pregnancy with the intent to answer some of these gaps in the literature.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11293775/

Record 350

pmcid: PMC11384347

pmid: 39262757

doi: 10.62347/ECMQ5753

title: Diagnostic potential of Shear wave elastography for central precocious puberty with breast development.

authors

- Liu L
- Zhou M
- Yao T
- Tan Y
- Liu J

journal: American journal of translational research

journalAbbr: Am J Transl Res

year: 2024

pubdate: 2024

volume: 16

issue: 8

pages: 3897-3906

abstract: PURPOSE: The aim of this study was to assess the accuracy and diagnostic use of shear wave elastography (SWE) in differentiating central precocious puberty (CPP) with breast development and to analyze the correlations between sex hormone levels and SWE parameters. METHODS: A total of 227 participants were included in this retrospective case-control study, including 113 girls with genuine precocious puberty breast development (the CPP group) and 114 with non-genuine precocious puberty breast development (the non-CPP group). The participants underwent clinical assessment, hormonal assays, and SWE using advanced ultrasound equipment. Statistical analyses, including t-tests, correlation analysis, logistic regression, and receiver operating characteristic (ROC) analysis, were performed to evaluate the diagnostic value of SWE and sex hormone levels

in differentiating CPP with breast development. RESULTS: There were no significant differences in clinical characteristics between the two groups. The sex hormone levels of estradiol, testosterone, luteinizing hormone (LH), follicle-stimulating hormone (FSH), and prolactin in the CPP group were significantly higher than those in the non-CPP group. Shear wave velocity (SWV) Maximum (Max), SWV Minimum (Min), SWV Mean, SWV standard deviation (SD), and SWV coefficient of variation in the CPP group were significantly higher than in the non-CPP group. Correlation analysis demonstrated significant positive correlations between LH, FSH, estradiol, and testosterone levels with various SWE parameters, indicating their clinical relevance. Logistic regression analysis identified substantial predictive potential of sex hormone levels and SWE parameters for genuine precocious puberty breast development. Additionally, the ROC analysis highlighted a high predictive value of the combined model of SWE parameters, with an area under the curve (AUC) of 0.903. CONCLUSION: The study underscores the correlations between sex hormone levels and SWE parameters. The superior predictive performance of the combined model of SWE parameters emphasizes the value of integrated SWE assessments for improving the accuracy of diagnosing genuine precocious puberty breast development.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11384347/

Record 351

pmcid: PMC11231073

pmid: 38983262

doi: 10.3389/fpubh.2024.1387976

title: Do inexperienced nurses in the lactation period experience workplace violence? A qualitative study.

authors

- Chen R
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- Wang D
- Wang Q
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journal: Frontiers in public health

journalAbbr: Front Public Health

year: 2024

pubdate: 2024

volume: 12

issue:

pages: 1387976

abstract: INTRODUCTION: Among clinical healthcare personnel, nurses face the highest proportion of workplace violence, which has a significant impact on their physical and mental well-being as well as their personal and professional lives. However, little is known about the effects of workplace violence on inexperienced breastfeeding nurses and their experiences during and after breastfeeding when they return to work. This study aimed to explore the experiences of inexperienced breastfeeding nurses who encountered workplace violence and its resulting impacts. METHODS: This study employed a descriptive qualitative design. Semi-structured in-depth interviews were conducted with 20 nurses working in various positions and departments at three tertiary hospitals. Purposive and maximum variation sampling techniques were employed. The interview data were analyzed using Colaizzi's method, and the research findings were reported according to Consolidated Criteria for Reporting Qualitative Studies (COREQ)standards. RESULTS: Inferences regarding workplace violence and risks for inexperienced breastfeeding nurses included physical labor (such as lifting heavy objects and performing cardiopulmonary resuscitation), conflicts, inadequate job skills, role confusion, occupational exposure risks, patient violence, and pressure from older adults. An inductive thematic investigation revealed the "Challenges faced during breastfeeding," "Conflicting professional and family roles," "Out of balance," and "Coping strategies." CONCLUSION: Inexperienced breastfeeding nurses experience

several negative consequences due to workplace violence. Therefore, it is essential to plan and implement preventive strategies and management programs that specifically target workplace violence among inexperienced breastfeeding nurses.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11231073/

Record 352

pmcid: PMC11402344

pmid: 39280118

doi: 10.2147/CLEP.S470959

title: Drug-Induced Gynecomastia: Data Mining and Analysis of the FDA Adverse Event Reporting System Database.

authors

- Yang X
- Zheng X
- Zhang M
- Huang J
- Huang P
- Wang J

journal: Clinical epidemiology

journalAbbr: Clin Epidemiol

year: 2024

pubdate: 2024

volume: 16

issue:

pages: 617-630

abstract: PURPOSE: Drug-induced gynecomastia significantly affects patient health and quality of life. This study aimed to perform an exploratory analysis of gynecomastia reports and the most commonly associated medications within the FAERS database. PATIENTS AND METHODS: A comprehensive analysis of the FAERS from January 2004 to December 2023 was conducted. Disproportionality analysis and subsequent sensitivity analysis were performed to identify drugs potentially associated with gynecomastia, utilizing the reported odds ratio (ROR). Logistic regression analysis was employed to assess potential risk factors. The Weibull shape parameter (WSP) test was used to assess the time-to-onset characteristics of the top drugs associated with gynecomastia. RESULTS: The study identified 30,265 cases of gynecomastia, primarily associated with nervous system drugs, accounting for 85.50% of cases. Notably, risperidone accounted for 80.81% of the total cases. Among the 165 agents with ≥ 5 cases of gynecomastia, the strongest signals were exhibited by risperidone (ROR 602.38, 95% CI 585.07-620.20), dutasteride (ROR 17.18, 95% CI 15.55-18.89), spironolactone (ROR 15.8, 95% CI 13.99-17.83), and paliperidone (ROR 7.16, 95% CI 6.55-7.84). In the sensitivity analysis of disproportionality, unexpected associations were observed, such as montelukast (n = 21, ROR 1.94, 95% CI 1.26-2.98). The logistic regression analysis indicated that the risk of risperidone-induced gynecomastia was significantly lower in adults compared to pediatric patients (OR 0.12, 95% CI 0.09-0.15) and in patients with higher body weight than in those with lower body weight (OR 5.24, 95% CI 3.62-7.76). The WSP test showed that gynecomastia induced by most of the top 10 common agents tends to occur in an early failure mode. CONCLUSION: The rankings and signal strengths of drugs associated with gynecomastia were extracted from the FAERS. The age distribution and time-to-onset distribution of the top 10 drugs linked to gynecomastia were investigated, which can facilitate accurate clinical recognition of drug-induced gynecomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11402344/

Record 353

pmcid: PMC11027302

pmid: 38331404

doi: 10.1093/bjr/tqae020

title: Early breast development in girls: the power of greyscale sonography and sonoelastography.

authors

- Keçeli M
- Akyürek N

journal: The British journal of radiology

journalAbbr: Br J Radiol

year: 2024

pubdate: 2024 Feb 28

volume: 97

issue: 1155

pages: 594-599

abstract: OBJECTIVE: Accurate distinction between central pubertal precociousness (PP) and premature thelarche (PT) is important to guide treatment. Both greyscale ultrasonography (US) and sonoelastography can be used to examine breast tissue. The aim of this study is to investigate the performance of breast US and strain elastographic (SE) in the diagnosis of increased breast volume in girls. METHODS: Sixty-three girls with breast development up to 8 years of age and diagnosed with PP and PT were included in the prospective study. Basal luteinizing hormone (LH), follicle-stimulating hormone (FSH), and estradiol (E2) values were obtained. Each bud was considered as a unit in US. Mediolateral (ML) and anteroposterior diameters (AP) were measured, and US grading was performed. Breast SE was examined, and strain index (SI) was calculated. US and laboratory findings were compared. RESULTS: Of the 121 buds examined, 39 (32.2%) were with PP (6.97 ± 2.44 years) and 82 (67.8%) were with PT (6.51 ± 2.52 years). Diameters were correlated with bone age, LH, FSH, and US grade. The mean AP showed a moderate difference in favour of PP between the groups (P < .06). The mean ML was higher in PP (P < .01). There was a difference in mean SI values (P < .004). Sensitivity and specificity were 71% and 61% for ML and 72% and 56% for SI, respectively. CONCLUSION: Both ML and US grading may help discriminate PP from PT. The role of sonoelastography requires further investigation. ADVANCES IN KNOWLEDGE: Ultrasound and SE show significant differences between PP and PT, but these are not sufficiently reliable to be of clinical use. The contribution of sonoelastography requires further study before it can be recommended. However, SI of breast tissue can be helpful in distinguishing PP and PT from other causes of early increase in breast volume.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11027302/

Record 354

pmcid: PMC11459112

pmid: 39312769

doi: 10.2196/60858

title: Effectiveness of 675-nm Wavelength Laser Therapy in the Treatment of Androgenetic Alopecia Among Indian Patients: Clinical Experimental Study.

authors

- Chandrashekar BS
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- Madeddu F
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journal: JMIR dermatology

journalAbbr: JMIR Dermatol

year: 2024

pubdate: 2024 Sep 23

volume: 7

issue:

pages: e60858

abstract: BACKGROUND: Androgenetic alopecia (AGA) is the most prevalent cause of hair loss around the world. OBJECTIVE: The purpose of this study was to evaluate the efficacy of laser stimulation with a 675-nm wavelength for the treatment of AGA in male and female Indian patients. METHODS: A total of 20 Indian healthy patients aged 23-57 years who presented a grade of alopecia stage I to stage V underwent one single pass with a 675-nm laser to the scalp area twice a week for a total of 8 sessions, followed by once a week for 4 sessions and once in 2 weeks for 2 sessions. There are 14 laser treatments in total. Macro- and dermatoscopic images have been acquired at T0 (baseline) and T1 (4 months). The vertex, frontal, and parietal areas of the scalp were evaluated. Many parameters were analyzed including hair count and hair density of terminal; mean thickness; vellus follicles; total follicular units; units with 1 hair, 2 hairs, 3 hairs, 4 hairs, and >4 hairs; unit density; and average hair/unit. RESULTS: The macroimages and dermatoscopic evaluations showed good improvement over the entire treated area, with a clear increase in the number of hairs and hair thickness. General parameters such as hair count and hair density showed a percentage increase of around 17%. The hair mean thickness parameters showed a significant (P<.001) percentage increase of 13.91%. Similar results were obtained for terminal and vellus hair: terminal hair count and hair density significantly (P=.04 and P=.01, respectively) increased by 17.45%, vellus hair count increased by 16.67% (P=.06), and the density of vellus hair increased by 16.61% (P=.06). CONCLUSIONS: The study findings demonstrate that the 675-nm laser system improved AGA in Indian patients, facilitating the anagen phase and improving hair density and other positive hair parameters.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11459112/

Record 355

pmcid: PMC11234817

pmid: 38982450

doi: 10.1186/s13023-024-03271-1

title: Efficacy and safety of D-penicillamine, trientine, and zinc in pediatric Wilson disease patients.

authors

- Lee EJ
- Woo MH
- Moon JS
- Ko JS

journal: Orphanet journal of rare diseases

journalAbbr: Orphanet J Rare Dis

year: 2024

pubdate: 2024 Jul 9

volume: 19

issue: 1

pages: 261

abstract: OBJECTIVES: Wilson disease (WD) is a rare genetic disease affecting copper metabolism and the biliary tract's copper excretion. Lifelong medication is necessary to prevent liver failure, neurological complications, and death. Although D-penicillamine (DPA), trientine, and zinc are used to treat WD, there is limited research on the long-term outcomes of these drugs, especially in children. This study aimed to evaluate the efficacy and safety of DPA, trientine, and zinc in patients diagnosed with WD during childhood. METHODS: Ninety out of 92 patients were included in the analysis, excluding two patients who underwent liver transplantation without drug treatment due to an acute liver failure diagnosis. Treatment outcomes and reasons for discontinuation of therapy in 148 treatment blocks (37 DPA, 50 trientine, and 61 zinc) were analyzed using Kaplan-Meier analysis. RESULTS: The median age at diagnosis was 8.3 years. There was a statistically significant difference in drug changes due to treatment ineffectiveness among the three drugs: trientine (22/50, 44%), zinc (15/61, 25%), and DPA (2/37, 5%) (all $p < 0.05$). Regarding drug changes due to adverse effects, the rate was the highest for DPA, followed by zinc and trientine. There were significant differences between DPA and zinc, zinc and trientine (all $p < 0.05$), but no significant difference was observed between DPA and zinc ($p = 0.22$). CONCLUSIONS: In pediatric WD, DPA, zinc, and trientine have therapeutic effects in that order. However, DPA and zinc are associated with more adverse effects compared to trientine.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11234817/

Record 356

pmcid: PMC11612006

pmid: 38958919

doi: 10.1007/s43032-024-01637-1

title: Efficacy of GnRH Pulses in Hypogonadism Secondary to Primary Empty Sella: Case Report.

authors

- Ran C
- Peng G
- Shen R
- Liao Q
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- Wang Q
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journal: Reproductive sciences (Thousand Oaks, Calif.)

journalAbbr: Reprod Sci

year: 2024

pubdate: 2024 Dec

volume: 31

issue: 12

pages: 3892-3898

abstract: This study aims to assess the effectiveness of pulsed gonadotropin-releasing hormone (GnRH) micropump replacement therapy in the treatment of hypogonadotropic hypogonadism (HH) caused by primary empty sella (PES).The efficacy of pulsed GnRH replacement therapy using the micropump was evaluated in a middle-aged male patient with HH who had experienced the loss of his only child. Relevant literature was also consulted to compare the differences between pulse GnRH treatment and conventional treatment in terms of the development of secondary sexual characteristics, sex hormone levels, sperm production rate, and sperm activity rate in male patient with HH.In this report, a 45-year-old male diagnosed with HH and PES presented with fatigue and decreased libido. The main characteristics included decreased follicle stimulating hormone (FSH)

levels of 0.03 mIU/mL, luteinizing hormone (LH) levels of 0.02 mIU/mL, and testosterone (T) levels of 0.72 nmol/L. Magnetic resonance imaging (MRI) revealed an empty sella. Semen analysis showed a small number of normal sperm with reduced motility. During treatment with the micropump pulse GnRH, the patient experienced no side effects and showed improvements in fatigue, reduced libido, sexual urge, anxiety, and feelings of inferiority. LH, FSH, and T levels returned to normal, while sperm activity rate increased to 79.9%. Ultimately, the patient's spouse achieved a natural pregnancy.Pulsed gonadotropin delivery using the micropump demonstrates good efficacy and tolerability, and aligns more closely with the physiological rhythm of GnRH secretion in the human body.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11612006/

Record 357

pmcid: PMC11721017

pmid: 39797240

doi: 10.3390/jcm14010158

title: Endocrine Hormones and Their Impact on Pubertal Gynecomastia.

authors

- Shi Z
- Xin M

journal: Journal of clinical medicine

journalAbbr: J Clin Med

year: 2024

pubdate: 2024 Dec 30

volume: 14

issue: 1

pages:

abstract: Pubertal gynecomastia (PG) is a common condition characterized by the abnormal development and hyperplasia of unilateral or bilateral breast tissue in adolescent males, affecting up to 50% of appropriately aged adolescents and exhibiting rising prevalence over recent years. The etiology of PG is multifaceted, encompassing physiological, pharmacological, and pathological factors. This narrative review synthesizes evidence from a comprehensive selection of peer-reviewed literature, including observational studies, clinical trials, systematic reviews, and case reports, to explore the pivotal role of endocrine hormones in the pathogenesis of PG. Specifically, it examines the effects of follicle-stimulating hormone (FSH), luteinizing hormone (LH), testosterone (T), estradiol (E2), progesterone (P), prolactin (PRL), growth hormone (GH), insulin-like growth factor-1 (IGF-1), thyroid hormones (T3, T4), parathyroid hormone (PTH), anti-Müllerian hormone (AMH), human chorionic gonadotropin (hCG), and leptin. By synthesizing current insights, this review underscores the intricate hormonal dynamics underlying PG and their implications for diagnosis and treatment. Conclusively, the findings advocate for a personalized approach in the clinical management of PG, with particular emphasis on the hormonal milieu as a cornerstone of therapeutic strategy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11721017/

Record 358

pmcid: PMC11182864

pmid: 38717607

doi: 10.1007/s00247-024-05934-4

title: Evaluating breast ultrasonography as a complementary diagnostic method in girls with central precocious puberty.

authors

- Bıyıklı E
- Helvacıoğlu D
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journal: Pediatric radiology

journalAbbr: Pediatr Radiol

year: 2024

pubdate: 2024 Jun

volume: 54

issue: 7

pages: 1156-1167

abstract: BACKGROUND: Assessment of breast development by physical examination can be difficult in the early stages and in overweight girls. OBJECTIVE: To investigate ultrasonography (US) for evaluation of early breast development. MATERIALS AND METHODS: In a prospective study, 125 girls (age 7.1 ± 1.5 years) with breast development before 8 years underwent US breast staging, breast volume, and elastography, in addition to clinical/hormonal evaluation for precocious puberty. Accuracy of US for determining breast development and predicting progression to central precocious puberty was investigated. RESULTS: Physical examination revealed glandular breast enlargement in 100 and predominantly lipomastia in 25. Breast US in the former confirmed glandular breast development in 92 (group 1, physical examination and US positive), but not in 8 (group 2, physical examination positive, US negative). Comparison of the two groups demonstrated lower Tanner and US staging, bone age/chronological age, basal luteinizing hormone (LH), breast volume, and uterine volume in group 2. In the 25 lipomastia patients, US demonstrated no breast tissue in 19 (group 3, physical examination and US negative), but US stage ≥ II in 6 (group 4, physical examination negative, US positive) without differences in clinical parameters. After follow-up of 19.8 ± 4.2 months, 46/125 subjects were diagnosed with precocious puberty. US stage, total breast volume, and shear-wave speeds were significantly higher in these 46 patients. Multivariate analyses demonstrated breast volume > 3.4 cc had odds ratio of 11.0, sensitivity of 62%, and specificity of 89, in predicting progression to precocious puberty, being second only to stimulated LH for all variables. CONCLUSION: Breast US is a useful predictive tool for diagnosis of precocious puberty in girls. Higher US stages and higher breast volume on US increased the likelihood of eventual diagnosis of precocious puberty.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11182864/

Record 359

pmcid: PMC11275570

pmid: 39061628

doi: 10.3390/diagnostics14141491

title: Evaluating Large Language Model (LLM) Performance on Established Breast Classification Systems.

authors

- Haider SA
- Pressman SM
- Borna S
- Gomez-Cabello CA

- Sehgal A
- Leibovich BC
- Forte AJ

journal: Diagnostics (Basel, Switzerland)

journalAbbr: Diagnostics (Basel)

year: 2024

pubdate: 2024 Jul 11

volume: 14

issue: 14

pages:

abstract: Medical researchers are increasingly utilizing advanced LLMs like ChatGPT-4 and Gemini to enhance diagnostic processes in the medical field. This research focuses on their ability to comprehend and apply complex medical classification systems for breast conditions, which can significantly aid plastic surgeons in making informed decisions for diagnosis and treatment, ultimately leading to improved patient outcomes. Fifty clinical scenarios were created to evaluate the classification accuracy of each LLM across five established breast-related classification systems. Scores from 0 to 2 were assigned to LLM responses to denote incorrect, partially correct, or completely correct classifications. Descriptive statistics were employed to compare the performances of ChatGPT-4 and Gemini. Gemini exhibited superior overall performance, achieving 98% accuracy compared to ChatGPT-4's 71%. While both models performed well in the Baker classification for capsular contracture and UTSW classification for gynecomastia, Gemini consistently outperformed ChatGPT-4 in other systems, such as the Fischer Grade Classification for gender-affirming mastectomy, Kajava Classification for ectopic breast tissue, and Regnault Classification for breast ptosis. With further development, integrating LLMs into plastic surgery practice will likely enhance diagnostic support and decision making.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11275570/

Record 360

pmcid: PMC11305854

pmid: 39111773

doi: 10.34763/jmotherandchild.20232701.d-24-00007

title: Evaluation of Maternal Risk Factors for Neonatal Hypernatremic Dehydration: A Systematic Review.

authors

- Maryam Z
- Forough R
- Ali M
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- Hassan B

journal: Journal of mother and child

journalAbbr: J Mother Child

year: 2024

pubdate: 2024 Feb 1

volume: 28

issue: 1

pages: 70-79

abstract: BACKGROUND: Neonatal hypernatremic dehydration (NHD) is a severe condition associated with

neonatal morbidity and mortality. PURPOSE: The present study evaluated maternal risk factors, including duration of maternal hospitalisation, primiparity, caesarean section, and pregnancy complications, as well as social factors, such as depression, fatigue, and inadequate support for NHD. DATA SOURCES: PubMed, Cochrane Library, International Scientific Indexing, Scopus, and Google Scholar were the databases searched until 2023. STUDY SELECTION: Articles written in English or Persian focusing on the relationship between maternal risk factors and NHD among neonates and providing sufficient information on NHD were included in this study. On the other hand, articles whose abstracts were only available were excluded. DATA EXTRACTION: The extracted data were presented in Excel software with the following titles: authors' names, year, type of study, study location, and maternal risk factors. The methodological quality of the articles was determined using the quality assurance tool for the diagnostic accuracy score (QUADAS). RESULTS: Of the 58 searched articles, 16 were investigated, which included five prospective, seven cross-sectional, and four retrospective articles. Maternal risk factors for NHD included labour and delivery complications, childbirth complications, factors causing insufficient breast milk intake (including breast milk insufficiency, nipple problems, wrong breastfeeding techniques, breast disorders, types of feeding, and breastfeeding training/counselling in pregnancy), as well as delivery and the postpartum period. IMPLICATIONS FOR PRACTICE AND RESEARCH: Maternal problems in pregnancy and delivery, breast disorders, breastfeeding status, maternal knowledge, and lactation skills are the most common maternal risk factors for NHD. Timely (antenatal) identification and proper management of maternal risk factors help reduce the incidence and severity of NHD complications.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11305854/

Record 361

pmcid: PMC11578192

pmid: 39568685

doi: 10.1097/GOX.0000000000006277

title: Evaluation of Traditional Liposuction, VASER Liposuction, and VASER Liposuction Combined with J-plasma in Management of Gynecomastia.

authors

- Ouf MAA
- Kishk TFAEH
- Elnahas MA

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2024

pubdate: 2024 Nov

volume: 12

issue: 11

pages: e6277

abstract: BACKGROUND: Because gynecomastia is one of the most considerable aesthetic procedures for men, we are researching in our article recent management methods for high grades of gynecomastia. METHODS: This prospective study included 45 individuals with gynecomastia, divided into 3 groups: group A (15 individuals) managed by traditional liposuction, group B (15 individuals) managed by vibration amplification of sound energy at resonance (VASER)-assisted lipoplasty, and group C (15 individuals) managed by VASER and J-plasma. Authors used a skin fold caliper for measurement of subareolar disk size and a Cutometer for assessment of skin pliability and firmness. The study was conducted at the Department of Plastic Surgery, Menoufia University Hospital, Egypt, and Reflect Center for Plastic Surgery in Riyadh, Saudi Arabia. RESULTS: Statistically significant differences were found among the groups in terms of reduction of subareolar disk size (assessed by skinfold caliper), skin redundancy (assessed by Cutometer), the need for a second stage of treatment, and patient satisfaction (by questionnaire). CONCLUSIONS: The combination of VASER and J-plasma is effective for managing grade 2 and 3 gynecomastia, resulting in significant reduction in subareolar

disk size, minimal skin redundancy with enhanced skin firmness, reduced need for further procedures, and improved patient satisfaction compared with traditional methods.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11578192/

Record 362

pmcid: PMC10905471

pmid: 38418148

doi: 10.21873/invivo.13492

title: External Volume Expansion: Timing and Effects on the Rate of Fat Graft Retention in BALB/c Nude Mice.

authors

- Chang YJ
- Kim YH
- Lee JS
- Ryu JY
- Choi KY
- Chung HY
- Cho BC
- Yang JD

journal: In vivo (Athens, Greece)

journalAbbr: In Vivo

year: 2024

pubdate: 2024 Mar-Apr

volume: 38

issue: 2

pages: 710-718

abstract: BACKGROUND/AIM: Fat grafting has been widely used for soft-tissue augmentation. External volume expansion (EVE) is a favorable tool for improvement in the rate of fat graft retention. However, few studies have focused on the most appropriate time for its implementation. In this study, BALB/c nude mice were used to investigate the effective time for the implementation of external volume expansion to improve the rate of fat retention. MATERIALS AND METHODS: Sixteen mice were divided into four groups, and EVE was performed at different time points before or both before and after fat grafting. Fat tissue from a human donor was injected into the mice following EVE. Visual assessment, micro-computed tomography analysis, and histopathological evaluation were used to assess fat retention. RESULTS: After 10 weeks, the group that underwent EVE 5 days before fat grafting demonstrated a significantly higher preserved fat volume, as determined by micro-computed tomography ($p<0.05$). Moreover, the group that received additional EVE after fat grafting exhibited a higher retention rate compared to the groups receiving EVE only before grafting ($p<0.05$). Histopathological analysis indicated that swelling, edema, and inflammation were more pronounced in the group with EVE immediately before grafting, while angiogenesis and lipogenesis were more active in the group with additional EVE after grafting. CONCLUSION: EVE is a safe and effective approach for improving the rate of fat graft retentions. Furthermore, the timing of external tissue expansion plays a crucial role in fat retention. Based on our animal study, performing EVE immediately before and after fat grafting may be an effective strategy for enhancing the rate of fat graft retentions.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10905471/

Record 363

pmcid: PMC11148562

pmid: 38642551

doi: 10.1016/j.xcrm.2024.101518

title: Genome-first evaluation with exome sequence and clinical data uncovers underdiagnosed genetic disorders in a large healthcare system.

authors

- Forrest IS
- Duffy Á
- Park JK
- Vy HMT
- Pasquale LR
- Nadkarni GN
- Cho JH
- Do R

journal: Cell reports. Medicine

journalAbbr: Cell Rep Med

year: 2024

pubdate: 2024 May 21

volume: 5

issue: 5

pages: 101518

abstract: Population-based genomic screening may help diagnose individuals with disease-risk variants. Here, we perform a genome-first evaluation for nine disorders in 29,039 participants with linked exome sequences and electronic health records (EHRs). We identify 614 individuals with 303 pathogenic/likely pathogenic or predicted loss-of-function (P/LP/LoF) variants, yielding 644 observations; 487 observations (76%) lack a corresponding clinical diagnosis in the EHR. Upon further investigation, 75 clinically undiagnosed observations (15%) have evidence of symptomatic untreated disease, including familial hypercholesterolemia (3 of 6 [50%] undiagnosed observations with disease evidence) and breast cancer (23 of 106 [22%]). These genetic findings enable targeted phenotyping that reveals new diagnoses in previously undiagnosed individuals. Disease yield is greater with variants in penetrant genes for which disease is observed in carriers in an independent cohort. The prevalence of P/LP/LoF variants exceeds that of clinical diagnoses, and some clinically undiagnosed carriers are discovered to have disease. These results highlight the potential of population-based genomic screening.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11148562/

Record 364

pmcid: PMC11296490

pmid: 39100734

doi: 10.29011/2574-7754.101700

title: Gestational Gigantomastia: Case Report and Review of Treatment Options.

authors

- Pleasant V
- Chapel E
- Momoh A
- Manorot A
- Carver A

journal: Annals of case reports

journalAbbr: Ann Case Rep

year: 2024

pubdate: 2024

volume: 9

issue: 2

pages:

abstract: Gestational gigantomastia is a rare and devastating condition involving rapid and excessive enlargement of the breast tissue during pregnancy or postpartum. We review the workup, assessment, and care of a patient with gestational gigantomastia with superimposed mastitis. Conservative treatment, medical management, and surgical options should be explored with patients. It is also important for providers to be cognizant of the psychological impact of this condition.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11296490/

Record 365

pmcid: PMC10912094

pmid: 38444922

doi: 10.1002/ccr3.8607

title: Gestational macromastia complicating pregnancy: A case report of unusual bilateral giant breasts in a patient who had no such problem in her previous pregnancies.

authors

- Lugata J
- Mrosso O
- Mchome B
- Mremi A

journal: Clinical case reports

journalAbbr: Clin Case Rep

year: 2024

pubdate: 2024 Mar

volume: 12

issue: 3

pages: e8607

abstract: KEY CLINICAL MESSAGE: Gestational macromastia is a rare disorder involving excessive breast tissue enlargement during pregnancy, potentially threatening the fetus and the mother. Our patient's previous pregnancies were more physiological, without any associated symptoms. Moderate bilateral enlargement of the breasts is a normal finding during pregnancy and lactation. Occasionally, there is a continuity from this physiological hypertrophy to massive breast hyperplasia or gigantomastia, causing complications that threaten the wellbeing of the fetus and the mother or raising alarm for a malignant disease. We present a case of a 28-year-old G3P2L2 with a gestational age of 29 weeks complaining of massive bilateral breast enlargement for 6 months, accompanied by a threatened abortion. The masses began gradually after conception and increased gradually over time. Notably, the breasts had been unremarkable in her two previous pregnancies. The breast ultrasonography and mammography findings pointed to a benign neoplasm. Histopathology of the lesions reported bilateral ductal hyperplasia. Conservative management and close follow-up were initiated. A cesarean section was performed due to cord prolapse, and a 1.3 kg male baby was extracted. Unfortunately, the neonate succumbed after 3 days due to apnea of prematurity. The patients' breast size subsided considerably with time.

Unusually large tumors can cause alarm for other pathologies, such as breast cancer. The radiological tests should reassure the attending practitioner, and the histological examination should confirm the diagnosis. An understanding of the typical and atypical clinico-pathologic characteristics of breast lesions occurring in pregnancy and lactation is essential for appropriate patient care.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10912094/

Record 366

pmcid: PMC11167359

pmid: 38815443

doi: 10.1016/j.ijscr.2024.109787

title: Giant breast lipoma: A case report.

authors

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journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2024

pubdate: 2024 Jul

volume: 120

issue:

pages: 109787

abstract: INTRODUCTION: Lipoma is the most common benign tumor of adipose tissue. Giant lipoma of the breast is defined as lesions larger than 10 cm and weighing more than 1000 g. A breast lipoma rapidly enlarging and fast growing; can be managed as a malignant tumor. It is crucial to make a correct diagnosis to prevent an overtreatment. CASE PRESENTATION: A 48-year-old patient presented with a painless, huge rapidly growing tumor in her right breast. Physical examination and imaging studies was suggestive of benign lipomatous breast tumor: A breast lipoma, a fibroadenolipoma or adenolipoma, an angiolipoma, or a breast fatty hamartoma. The patient underwent surgical excision of the mass, and histological examination confirmed the diagnosis of a giant breast lipoma. DISCUSSION: Giant breast lipoma is a rare benign tumor that develops in the breast tissue. They can mimic various breast conditions, even neoplastic conditions. Giant breast lipomas are often treated with surgical excision to avoid recurrence. CONCLUSION: Giant breast lipoma rapidly growing can pose a diagnostic challenge due to its resemblance to various benign or malignant pathologies. Unnecessary invasive investigations can be avoided with better understanding and improved imaging-based diagnosis of giant breast lipoma.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11167359/

Record 367

pmcid: PMC11110870

pmid: 38769628

doi: 10.1080/13880209.2024.2350233

title: Guidelines for the clinical application of the Xihuang pill for the prevention and treatment of breast hyperplasia diseases.

authors

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- Lin H
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- Li L
- Liu T
- Ouyang Y
- Lei K
- Chen J
- Peng W
- Qiu Z
- Cai Q
- Liang Q
- Liu C
- Li Y
- Zhu L
- Zhang Z
- Liu X
- Lin L
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journal: Pharmaceutical biology

journalAbbr: Pharm Biol

year: 2024

pubdate: 2024 Dec

volume: 62

issue: 1

pages: 472-479

abstract: CONTEXT: The Xihuang pill (XHP) is a traditional Chinese medicine formulation that has been historically used in the prevention and treatment of proliferative breast diseases. However, there is a lack of guidelines that offer recommendations for its clinical use. OBJECTIVE: The task force from the Chinese Guangdong Pharmaceutical Association aims to develop evidence-based guidelines for XHP to prevent and treat proliferative breast diseases. METHODS: We searched six Chinese and English electronic databases, including the China National Knowledge Infrastructure, the Chinese Scientific Journal Database, the Wanfang

Medical Database, PubMed, and Embase, up to November 1, 2022. Publications (case reports, clinical observation, clinical trials, reviews) on using XHP to treat proliferative breast diseases were manually searched. The search terms were Xihuang pill, hyperplasia of the mammary gland, breast lump, and mastalgia. The writing team developed recommendations based on the best available evidence. RESULTS: Treatment should be customized based on syndrome identification. We recommend using XHP for the prevention and treatment of breast hyperplasia disease when a patient presents the following syndromes: concurrent blood stasis syndrome, concurrent phlegm-stasis syndrome, and concurrent liver fire syndrome. Safety indicators, including blood analysis and liver and kidney function monitoring, should be performed regularly during treatment. CONCLUSIONS: Current clinical evidence suggests that XHP can be used as a standalone treatment or in conjunction with other medications to prevent and manage breast hyperplasia diseases. More randomized controlled studies are warranted to establish high-quality evidence of its use.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11110870/

Record 368

pmcid: PMC11076233

pmid: 38712491

doi: 10.6065/apem.2346142.071

title: Gynecomastia in adolescent males: current understanding of its etiology, pathophysiology, diagnosis, and treatment.

authors

- Metwalley KA
- Farghaly HS

journal: Annals of pediatric endocrinology & metabolism

journalAbbr: Ann Pediatr Endocrinol Metab

year: 2024

pubdate: 2024 Apr

volume: 29

issue: 2

pages: 75-81

abstract: Gynecomastia (GM) is a common and continuously evolving condition that commonly occurs during adolescence. It is the source of significant embarrassment and psychological stress in adolescent males. GM is characterized by enlargement of the male breast due to the proliferation of glandular ducts and stromal components. The main cause of GM during adolescence is physiological or pubertal GM, which is primarily attributed to an imbalance between estrogen and androgen activity. Physiological GM is typically transient and resolves within several months, although it may take several years to resolve. GM may also be caused by other pathological conditions and could be indicative of an endocrine disease. It is crucial to understand the pathogenesis of GM to distinguish it from normal developmental variants due to pathological causes. The aim of this review is to highlight the significance of GM during adolescence in terms of potential etiologies, clinical and laboratory diagnoses, and current management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11076233/

Record 369

pmcid: PMC10832307

pmid: 38304650

doi: 10.7759/cureus.51509

title: Gynecomastia on Thoracic Computed Tomography.

authors

- Deniz MA
- Matsar Öz R

journal: Cureus

journalAbbr: Cureus

year: 2024

pubdate: 2024 Jan

volume: 16

issue: 1

pages: e51509

abstract: Background and objective Gynecomastia is a benign proliferation of ductal epithelium in the retroareolar region in male patients. The aim of this study was to investigate the frequency of gynecomastia in male patients who underwent thoracic computed tomography (CT) imaging at our clinic, assess possible causes, highlight the imaging characteristics of gynecomastia, and compare our findings with the literature. Materials and methods Male patients over 18 years of age who underwent thoracic CT imaging in our clinic were included in the study. Patients were initially assessed based on age and the presence of gynecomastia. The patients with gynecomastia were evaluated in terms of age, gynecomastia localization (right, left, and bilateral), gynecomastia type (nodular, dentritic, and diffuse), and possible etiology. Results The study included 1500 patients with a mean age of 45.6±21.7 years, and 470 (31.3%) patients had gynecomastia. Gynecomastia was on the right side in 11.3%, on the left side in 11.1%, and bilateral in 77.7% of the patients. Gynecomastia was nodular in 52.1%, dendritic in 35.3%, and diffuse in 17.2% of the patients. The causative factor could not be identified in 44.3% of the patients with gynecomastia. Among cases where the etiology was identified (56.7%), the most common factors were cancer (23.4%), chronic kidney disease (CKD) (13.2%), and chronic hepatitis B (10.7%). Conclusion When evaluating thoracic CT, the breast area, in addition to the lungs, chest wall, and bone structures, should also be evaluated carefully. With the increased use of thoracic CT scans, incidentally detected gynecomastia in patients is also on the rise. Knowing the presence of gynecomastia is very important for the clinician to determine the etiology and treat the underlying disease. Therefore, detecting and reporting gynecomastia on thoracic CT can prevent unnecessary advanced breast imaging methods and play a very important role in treating the underlying etiology.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10832307/

Record 370

pmcid: PMC11873591

pmid: 38808551

doi: 10.7555/JBR.37.20230310

title: hUCMSC-derived extracellular vesicles relieve cisplatin-induced granulosa cell apoptosis in mice by transferring anti-apoptotic miRNAs.

authors

- Tang W
- Yan H
- Chen X
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journal: Journal of biomedical research

journalAbbr: J Biomed Res

year: 2024

pubdate: 2024 May 29

volume: 39

issue: 1

pages: 36-49

abstract: Premature ovarian insufficiency (POI) caused by chemotherapy is a common complication in female cancer survivors of childbearing age. Traditional methods, including mesenchymal stem cell (MSC) transplant and hormone replacement therapy, have limited clinical application because of their drawbacks, and more methods need to be developed. In the current study, the potential effects and underlying mechanisms of human umbilical cord MSC-derived extracellular vesicles (hUCMSC-EVs) were investigated in a cisplatin (CDDP)-induced POI mouse model and a human granulosa cell (GC) line. The results showed that hUCMSC-EVs significantly attenuated body weight loss, ovarian weight loss, ovary atrophy, and follicle loss in moderate-dose (1.5 mg/kg) CDDP-induced POI mice, similar to the effects observed with hUCMSCs. We further found that the hUCMSC-EVs inhibited CDDP-induced ovarian GC apoptosis by upregulating anti-apoptotic miRNA levels in GCs, thereby downregulating the mRNA levels of multiple pro-apoptotic genes. In general, our findings indicate that the moderate-dose chemotherapy may be a better choice for clinical oncotherapy, considering effective rescue of the oncotherapy-induced ovarian damage with hUCMSC-EVs. Additionally, multiple miRNAs in hUCMSC-EVs may potentially be used to inhibit the chemotherapy-induced ovarian GC apoptosis, thereby restoring ovarian function and improving the life quality of female cancer patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11873591/

Record 371

pmcid: PMC11242149

pmid: 38999285

doi: 10.3390/jcm13133719

title: Illegal Use of Testosterone and Other Anabolic-Androgenic Steroids in the Population of Amateur Athletes in Wrocław, Poland-An Unfavorable Lifestyle Trend in the Population of Men of Reproductive Age.

authors

- Skrzypiec-Spring M
- Pokrywka A
- Bombała W
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journal: Journal of clinical medicine

journalAbbr: J Clin Med

year: 2024

pubdate: 2024 Jun 26

volume: 13

issue: 13

pages:

abstract: Background: One factor that may negatively impact male reproductive health is the illegal use of testosterone and anabolic-androgenic steroids. This study aimed to evaluate the prevalence of testosterone use in recreational athletes, as well as factors associated with its use, and to determine the profile of a person using testosterone. Methods: A cross-sectional analysis of data from an anonymous, online questionnaire of men recruited from gyms, randomly selected in Wrocław, Poland, has been performed. The minimal sample size was evaluated with the univariate logistic regression model. The association between testosterone use and other factors was also evaluated with the univariate logistic regression model. Results: A total of 35% of respondents used testosterone. The main purposes of testosterone use were the improvement of training effects and the improvement of body shape. The respondents most likely to use testosterone and other anabolic-androgenic steroids were men aged 26-35, whose earnings were at the level of the middle class or higher, who were married, had children, had training experience of at least 6 months, exercised at least once a week, took part in weightlifting competitions, were managers in a corporation or enterprise, or were self-employed. Most of the people using testosterone had self-treated side effects. Conclusions: The profile of the person most likely to use testosterone corresponds to the characteristics of men in optimal socio-demographic conditions for reproduction. These results indicate that this is a significant social problem that may impact male reproductive health.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11242149/

Record 372

pmcid: PMC11412099

pmid: 39301253

doi: 10.3892/etm.2024.12714

title: Imatinib-induced gynecomastia: A case report.

authors

- Li XL
- Li M
- Tu SK
- Fan HJ
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- Tian J
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journal: Experimental and therapeutic medicine

journalAbbr: Exp Ther Med

year: 2024

pubdate: 2024 Nov

volume: 28

issue: 5

pages: 425

abstract: Chronic myeloid leukemia is a myeloproliferative neoplasm characterized by the unregulated and abnormal proliferation of both mature and immature granulocytes, which results in the proliferation of peripheral blood leukocytes. Imatinib, a tyrosine kinase inhibitor, is the first-line treatment for patients diagnosed with chronic myeloid leukemia. However, despite its favorable safety profile, imatinib use is associated with a number of side effects. Gynecomastia is a rare adverse effect of imatinib treatment and may be associated with an imbalance in sex hormones. The present study reports the case of a patient with chronic myeloid leukemia diagnosed with gynecomastia after imatinib treatment. The aim of the present report was to highlight to

clinicians this adverse reaction to imatinib treatment and investigate a treatment strategy with fewer side effects.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11412099/

Record 373

pmcid: PMC11087426

pmid: 38151679

doi: 10.1007/s40199-023-00501-y

title: Imiquimod as a new treatment in refractory idiopathic granulomatous mastitis: report of two cases.

authors

- Alipour S
- Gholami B
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journal: Daru : journal of Faculty of Pharmacy, Tehran University of Medical Sciences

journalAbbr: Daru

year: 2024

pubdate: 2024 Jun

volume: 32

issue: 1

pages: 443-447

abstract: INTRODUCTION: Idiopathic granulomatous mastitis (IGM) is a rare chronic inflammatory lesion of the breast that mimics breast cancer or infection. Immunological pathogenesis is strongly suggested for the disease. REASON FOR THE REPORT: The treatment remains controversial, comprising a spectrum from observation or NSAIDs to immunosuppressive agents and surgery. Intractable cases are not uncommon and represent a major treatment challenge. Therefore in this study, we examine the effect of a topical immunomodulator agent, imiquimod, on refractory IGM. Patient 1 had IGM for 9 months and had not responded to the existing treatments. She responded to a 7-week course of imiquimod. In patient 2, the disease had begun 4 months sooner and had been resistant to all treatments; it responded to imiquimod after 4 weeks. Ulcers appeared on the skin of both patients but resolved safely. OUTCOME: Both patients were very satisfied with the results. Imiquimod can be an appropriate local treatment with limited adverse effects in refractory IGM. We propose similar studies to assess the efficacy of imiquimod in IGM further, paying attention to the possibility of developing skin wounds.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11087426/

Record 374

pmcid: PMC11012914

pmid: 38610604

doi: 10.3390/jcm13071839

title: Impact of Pre-Gestational BMI and Gestational Weight Gain on Fetal Development Outcomes in Adolescent Pregnant Women.

authors

- Grobeisen-Duque O
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- Zaga-Clavellina V
- Aguilera-Arreola MG
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journal: Journal of clinical medicine

journalAbbr: J Clin Med

year: 2024

pubdate: 2024 Mar 22

volume: 13

issue: 7

pages:

abstract: Background. Gestational weight gain (GWG) constitutes an essential aspect of the gestational process. Due to factors such as pregestational body mass index (BMI), nutritional intake, level of physical activity, and psychological aspects, the recommended GWG may not be achieved, leading to adverse neonatal outcomes. Adolescents, due to their physiological and mental developmental stage, are at a higher risk of inappropriate GWG. Our aim is to highlight the importance of GWG in our population and to determine the correlation with perinatal outcomes. Methods. Pregnant adolescents who attended a tertiary care institution for prenatal care were included; maternal data such as preBMI and GWG were used to determine maternal and neonatal outcomes using the chi-square test and OR determination. Results. A total of 202 adolescent pregnant patients were included, comprising those with inadequate GWG (n = 70), adequate GWG (n = 85), and excessive GWG (n = 47). A statistically significant association was found between low BMI and inadequate GWG. Patients with inadequate GWG demonstrated a correlation with IUGR and low birth weight, while patients with excessive GWG gave birth to macrosomic neonates. Conclusion. We concluded that previous habits play a significant role in determining weight gain throughout pregnancy. GWG has a direct impact on neonatal growth and development.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11012914/

Record 375

pmcid: PMC11175848

pmid: 38875389

doi: 10.1097/MD.0000000000038514

title: Importance of parental anxiety in management of developmental breast conditions in children: A study with a prospective hierarchical regression model.

authors

- Özdamar MY
- Biçer Ş

journal: Medicine

journalAbbr: Medicine (Baltimore)

year: 2024

pubdate: 2024 Jun 14

volume: 103

issue: 24

pages: e38514

abstract: The parent is the most critical link and decision-maker between the patient and the healthcare provider in treating many pediatric diseases. This entity is essential for the management of pediatric breast diseases for which the rate of surgical intervention is known to be very low. Although previous publications have emphasized that pediatric breast diseases may cause alarming anxiety in parents, the demographic factors that influence this anxiety have not been investigated. Even if practitioners complete patient management with appropriate procedures, treatment is incomplete if the questions remain unanswered. In this observational prospective study, we investigated the demographic factors that affect parental anxiety, which should be prioritized to prevent incomplete management. The Beck Anxiety Inventory score (BAS) created by the parents of 409 boys and girls aged 0 to 17 with breast conditions was recorded at the diagnosis, termination of treatment, and final control stages. A 2-stage hierarchical logistic regression model was applied to show how strongly the demographic characteristics of parents and their children predicted the parental BAS. Of the demographic characteristics, there was a significant correlation ($P < .05$) between the patient's sex, age, developmental period, Tanner stage, referral status, management method, family's place of residence, economic distress, and BAS. However, according to the 2-stage hierarchical regression model, only 3 demographic characteristics, the patient's gender, place of residence, and method used in patient management, significantly predicted BAS ($P < .05$, $\Delta R2 = .35$). Among the many factors that affect anxiety experienced by parents whose son or daughter has breast problems, the gender of the child, place of residence of the family, and management methods used by the practitioner are demographic characteristics that should be taken into consideration.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11175848/

Record 376

pmcid: PMC11342634

pmid: 39175092

doi: 10.1186/s13006-024-00666-5

title: Incidence and factors influencing delayed onset of lactation: a systematic review and meta-analysis.

authors

- Peng Y
- Zhuang K
- Huang Y

journal: International breastfeeding journal

journalAbbr: Int Breastfeed J

year: 2024

pubdate: 2024 Aug 22

volume: 19

issue: 1

pages: 59

abstract: BACKGROUND: Breastfeeding has many benefits for mothers and infants. Lactogenesis II is one of the key steps in the implementation of breastfeeding. If lactogenesis II occurs more than 72 h after delivery, it is termed delayed onset of lactation (DOL). DOL is associated with decreased milk production, shortened breastfeeding time, and pathological neonatal weight loss. A comprehensive summary of the incidence and factors influencing DOL is needed to provide a basis for improving breastfeeding practices and health outcomes. METHODS: Studies on the incidence and factors influencing DOL were retrieved from 13 Chinese and English databases (PubMed, Embase, Web of Science, Cochrane Library, CINAHL, etc.) from database inception to August 2023. Two researchers independently conducted the study screening, data extraction and quality evaluation. Stata 16.0 SE software was used for data analysis, and sensitivity analysis and publication bias tests were also performed. The qualitative description method was used to analyse studies that could not be

combined quantitatively. RESULTS: A total of 35 studies involving 19,176 parturients, including 4,922 who had DOL, were included. The mean Newcastle−Ottawa scale score of the included studies was ≥ 6, indicating that the quality was relatively high. Finally, the incidence of DOL was 30%, and 13 factors influencing DOL with robust results and no publication bias were obtained: prepregnancy body mass index (overweight or obesity), gestational diabetes, gestational hypertension, thyroid disease during pregnancy, serum albumin levels (< 35 g/L), parity, (unscheduled) caesarean section, caesarean section history, daily sleep duration, gestational age, birth weight (< 2.5 kg), breastfeeding guidance and daily breastfeeding frequency. However, there were still six influencing factors with undetermined associations: age, gestational weight gain, birth weight (≥ 4 kg), anxiety, time of first breastfeeding session (maternal separation) and breast massage or treatment. CONCLUSIONS: The incidence of DOL is high. Clinicians should pay attention to parturients at high risk of DOL and formulate targeted prevention strategies according to the influencing factors to reduce the occurrence of DOL and promote better maternal and infant outcomes. TRIAL REGISTRATION: PROSPERO (ID: CRD42023458786), September 10, 2023.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11342634/

Record 377

pmcid: PMC11471039

pmid: 39398702

doi: 10.7759/cureus.69323

title: Incidence of Hematoma Following Breast Reduction in Patients Taking Selective Serotonin Reuptake Inhibitor or Serotonin-Norepinephrine Reuptake Inhibitor: A Retrospective Review.

authors

- Duncan A
- Stewart K
- Dow T
- Williams J

journal: Cureus

journalAbbr: Cureus

year: 2024

pubdate: 2024 Sep

volume: 16

issue: 9

pages: e69323

abstract: Background Selective serotonin reuptake inhibitor (SSRI) and serotonin-norepinephrine reuptake inhibitor (SNRI) use is more common in the plastic surgery population compared to the general population. This study was designed to assess the theoretical effect of SSRIs and SNRIs on platelet function and the potential for increased bleeding risk. This study sought to establish the incidence of postoperative bleeding following routine bilateral breast reduction for patients on SSRIs or SNRIs. The outcomes of this study contribute to the discussion of whether these medications should be discontinued before elective surgery. Methodology A retrospective chart review of all patients who received bilateral breast reduction surgery over a 10-year period was performed. Patient charts were reviewed for postoperative hematoma formation as well as medications being used around the time of surgery. The rate of hematoma formation in patients actively taking SSRIs or SNRIs at the time of surgery was compared with the rest of the study population. Results A total of 1,022 patients met the inclusion criteria for the study. The overall incidence of postoperative hematoma was 7.7%. Of these, 1.9% of patients had clinically significant hematomas that required operative evacuation, and the remaining were treated conservatively. The only variable associated with a significantly higher risk of hematoma formation was advanced age (p = 0.005). Conclusions There was no significant difference in hematoma incidence after breast reduction in patients taking SNRIs or SSRIs compared with the general population. This

contradicts some of the previously published literature and can hopefully guide clinicians in counseling their patients preoperatively.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11471039/

Record 378

pmcid: PMC11632603

pmid: 39664084

doi: 10.22551/2024.45.1104.10300

title: Innovative surgical management of gestational gigantomastia.

authors

- Abdulkarim S
- Aldien AS
- Alazzam A
- Martel K

journal: Archive of clinical cases

journalAbbr: Arch Clin Cases

year: 2024

pubdate: 2024

volume: 11

issue: 4

pages: 110-113

abstract: Gigantomastia is a rare condition characterized by excessive breast enlargement, which can lead to physical and psychological distress. Gestational gigantomastia (GG) occurs during pregnancy, often presenting significant management challenges. This case contributes to the limited literature on GG management by highlighting the successful use of the Goldilocks technique combined with free nipple grafting, offering insights into an effective surgical approach. A pregnant woman presented with severe GG. She underwent bilateral skin-sparing mastectomy and immediate reconstruction using the Goldilocks technique with free nipple grafting. The intervention provided both functional and aesthetic outcomes, significantly improving the patient's quality of life. This case underscores the effectiveness of modern reconstructive surgical techniques in managing GG, particularly in complex cases where hormonal therapy is insufficient.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11632603/

Record 379

pmcid: PMC11231714

pmid: 38901382

doi: 10.1016/j.ijscr.2024.109873

title: Innovative technique for managing extreme relapsing bilateral pseudoangiomatous stromal hyperplasia (PASH) in a young woman: A case report highlighting a novel intervention in reconstruction.

authors

- Pakbaz Y
- Hoseinpour P
- Olamaeian F
- Nafissi N

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2024

pubdate: 2024 Jul

volume: 120

issue:

pages: 109873

abstract: INTRODUCTION: Pseudoangiomatous stromal hyperplasia (PASH) is a rare breast stromal lesion that typically manifests clinically as a palpable unilateral, painless lump that is freely movable and has a rubbery or firm consistency. The diagnosis can be confirmed by a core needle biopsy (CNB) or surgical excision. Treatment options include medical treatment with hormonal management for asymptomatic patients or local excision and breast reduction in rare cases. CASE PRESENTATION: We reported the case of a 24-year-old woman with a history of taking contraceptive pills for about a year. Examination revealed extremely enlarged, sore, and swollen breasts, particularly the right side, marking her third PASH relapse. The patient underwent a surgical skin-reducing mastectomy (SRM) using a novel technique with an immediate prepectoral implant covered by a dermal flap to reconstruct the breast shape due to the large PASH lesions and aiming for optimal cosmetic outcomes. The procedure was complication-free with no recurrence after 18 months of follow-up. DISCUSSION: Mastectomy followed by immediate implantation offers benefits such as prompt restoration of breast shape with fewer surgeries. CONCLUSION: This case report highlights the successful use of immediate implantation in reconstructing large recurrent benign breast diseases. The outcomes indicate that immediate implantation shows promise as a suitable choice for carefully selected patients managing large, relapsing bilateral benign breast diseases. However, due to common complications such as infection and implant loss, it is not generally recommended for benign lesions. The decision should be made on a case-by-case basis, considering the size, recurrence, and individual suitability.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11231714/

Record 380

pmcid: PMC10826571

pmid: 38228396

doi: 10.1136/bmjopen-2023-076608

title: Is male gynaecomastia associated with an increased risk of death? A nationwide register-based cohort study.

authors

- Bräuner EV
- Uldbjerg C
- Lim YH
- Beck A
- Hueg T
- Juul A

journal: BMJ open

journalAbbr: BMJ Open

year: 2024

pubdate: 2024 Jan 16

volume: 14

issue: 2

pages: e076608

abstract: OBJECTIVE: Recent evidence supports that gynaecomastia may predict long-term morbidity, but evidence on the association with death and causes of death in males with gynaecomastia is lacking. The objective of this work is to estimate the risk of death in men diagnosed with gynaecomastia and evaluate whether this was conditional on underlying aetiologies of gynaecomastia. DESIGN: A nationwide register-based cohort study. SETTING: Nationwide Danish national health registries. PARTICIPANTS: Males were diagnosed with incident gynaecomastia (n=23 429) from 1 January 1995 to 30 June 2021, and each was age and calendar matched to five randomly population-based males without gynaecomastia (n=117 145). INTERVENTIONS: Not applicable. PRIMARY AND SECONDARY OUTCOMES: Gynaecomastia was distinguished between males without (idiopathic) and males with a known pre-existing risk factor. Cox regression models and Kaplan-Meier analyses estimated associations between gynaecomastia and death (all cause/cause specific). RESULTS: We identified a total of 16 253 males with idiopathic gynaecomastia and 7176 with gynaecomastia and a known pre-existing risk factor . Of these, 1093 (6.7%) and 1501 (20.9%) died during follow-up, respectively. We detected a 37% increased risk of all-cause death in males with gynaecomastia in the entire cohort (HR 1.37; 95% CI 1.31 to 1.43). Death risk was highest in males diagnosed with gynaecomastia and a known pre-existing risk factor (HR 1.75; 95% CI 1.64 to 1.86) compared with males with idiopathic gynaecomastia (HR 1.05; 95% CI 0.98 to 1.13). Specific causes of increased death were malignant neoplasms and circulatory, pulmonary and gastrointestinal diseases. Of the latter, an over fivefold risk of death from liver disease was detected (HR 5.05; 95% CI 3.97 to 6.42). CONCLUSIONS: Males diagnosed with gynaecomastia are at higher risk of death, observed mainly in males with a known pre-existing risk factor of gynaecomastia. These findings will hopefully stimulate more awareness among healthcare providers to potentially apply interventions that aid in alleviating underlying risk factors in males with this condition.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10826571/

Record 381

pmcid: PMC10762326

pmid: 38173893

doi: 10.1002/ccr3.8398

title: Large tumoral pseudoangiomatous stromal hyperplasia with ER/PR stromal negativity in a 20-year-old female: A rare case report.

authors

- Mansour S
- Abdul Rahman SA
- Kazour A
- Salama I
- Shmayes H
- Rajab S
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journal: Clinical case reports

journalAbbr: Clin Case Rep

year: 2024

pubdate: 2024 Jan

volume: 12

issue: 1

pages: e8398

abstract: Pseudoangiomatous stromal hyperplasia (PASH) is a rare lesion of the breast stromal tissue with unknown mechanism. Hormonal stimulation of mammary myofibroblasts is the most important theory due to stromal positivity of progesterone receptor (PR) or/and estrogen receptor (ER). We report a case of PASH with

stromal PR/ER negativity.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10762326/

Record 382

pmcid: PMC11155343

pmid: 38846510

doi:

title: Late Hematoma After Feminizing Augmentation Mammoplasty Mimicking Breast Implant-Associated Anaplastic Large Cell Lymphoma (BIA-ALCL).

authors

- Ueno K
- Sakata Y
- Kawaji M
- Miyasaka M
- Asamura S

journal: Eplasty

journalAbbr: Eplasty

year: 2024

pubdate: 2024

volume: 24

issue:

pages: e32

abstract: We report the case of an Asian transgender patient with late hematoma after feminizing mammoplasty. Bilateral silicone breast implants were inserted into the patient 25 years previously. The right breast gradually became swollen without any specific cause, along with erythema and pain. Positron emission tomography showed right axillary lymphadenopathy. The mass and the axillary lymph node were surgically removed. Pathologic examination of the excised specimen revealed only hematoma formation and inflammatory granulation. At follow-up at 6 months postoperatively there was no reformation of hematoma. The presented symptoms are similar to those of breast implant-associated anaplastic large cell lymphoma, so there can be difficulty in differentiating between these 2 complications. We compared the clinical characteristics between our case of late hematoma and reported breast implant-associated anaplastic large cell lymphoma after feminizing mammoplasty. Life-threatening breast implant-associated anaplastic large cell lymphoma should be ruled out from late hematoma according to the National Comprehensive Cancer Network screening guidelines.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11155343/

Record 383

pmcid: PMC11679183

pmid: 39734375

doi: 10.1055/s-0044-1789005

title: Long-Term Outcome of Surgery for Grade 4 Gynecomastia: A Single-Center Experience.

authors

- Goel A
- Punia S
- Gupta A

journal: Indian journal of plastic surgery : official publication of the Association of Plastic Surgeons of India

journalAbbr: Indian J Plast Surg

year: 2024

pubdate: 2024 Dec

volume: 57

issue: 6

pages: 455-460

abstract: Background Gynecomastia results in a feminine appearance of the male chest, leading to social embarrassment and loss of self-esteem in the afflicted males. Grade 4 gynecomastia is expected to have less than perfect results with liposuction and gland excision alone. This study was done to assess the long-term outcome of this surgery for grade 4 gynecomastia. Materials and Methods From January 2021 to December 2022, 81 patients with grade 4 gynecomastia were treated by us. All the patients underwent vibration amplification of sound energy at resonance (VASER) and suction-assisted liposuction of the chest and side rolls with excision of the gland with crescentic lift in the cases with ptosis. A retrospective study was done to analyze the long-term surgical outcomes in these patients by review of clinical records. Results Symmetry was achieved in 37/39 patients with grade 4a gynecomastia but only in 33/42 patients with grade 4b gynecomastia. The inframammary fold disappeared in 35/39 patients with grade 4a gynecomastia but only in 25/42 of grade 4b gynecomastia patients. Ptosis was corrected in 35/42 grade 4b gynecomastia patients. The mean follow-up was 15 months (range: 12-24 months). Only seven patients desired a second stage to correct the remaining deformity. Conclusion Liposuction with gland removal alone in grade 4a gynecomastia and with liposuction with crescentic nipple-areola complex (NAC) lift in patients of grade 4b gynecomastia give satisfactory results in patients with massively enlarged breasts. While grade 4a gynecomastia has overall better results and lesser complications as compared with grade 4b gynecomastia, the latter also has acceptable outcomes. Realistic prognosis needs to be explained to the patient preoperatively.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11679183/

Record 384

pmcid: PMC11021050

pmid: 38633681

doi: 10.25259/IJPC_165_2023

title: Management of Lymphoedema and Lymphorrhoea with Wrap around Compression in Breast Secondary to Carcinoma Breast: A Case Report.

authors

- Shukla A
- Gonnade N
- Kamal M

journal: Indian journal of palliative care

journalAbbr: Indian J Palliat Care

year: 2024

pubdate: 2024 Jan-Mar

volume: 30

issue: 1

pages: 85-88

abstract: Lymphoedema secondary to breast cancer is one of the most unfortunate outcomes, and lymphoedema secondary to breast cancer in the ipsilateral upper extremity and its management, has been

described in the literature. We are reporting a case of lymphoedema and lymphorrhoea in the breast itself, managed by compression bandaging with limited resources and the use of active pharmacological agents in the palliative care unit of a tertiary care hospital. Lymphoedema and lymphorrhoea in the breast are not very common to see in clinics, but management of this case even though no protocol available, was done by modification of bandaging techniques available for limb lymphoedema management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11021050/

Record 385

pmcid: PMC11179057

pmid: 38881950

doi: 10.1016/j.jcte.2024.100355

title: Maximizing surgical outcomes with gender affirming hormone therapy in gender affirmation surgery.

authors

- Gelles-Soto D
- Ward D
- Florio T
- Kouzounis K
- Salgado CJ

journal: Journal of clinical & translational endocrinology

journalAbbr: J Clin Transl Endocrinol

year: 2024

pubdate: 2024 Jun

volume: 36

issue:

pages: 100355

abstract: Gender Affirmation Surgery (GAS) is a super specialized subset within the field of plastic and reconstructive surgery (PRS) that is ever evolving and of increasing interest to the PRS community. It is a multifaceted process which, in addition to surgical therapy, involves mental health therapy and hormonal therapy. One rapidly emerging interest within GAS is the role that gender affirming hormone therapy (GAHT) plays in enhancing surgical outcomes. GAHT has been used adjunctively with GAS as a comprehensive therapy to ameliorate gender dysphoria. This literature review will examine the positive effects of GAHT on the surgical outcomes on GAS, as well as other important considerations prior to surgery. As such, the primary objective of this literature review is to evaluate and assess the current evidence concerning the efficacy and safety of GAHT, as it relates to Gender Affirmation Surgery procedures.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11179057/

Record 386

pmcid: PMC11588962

pmid: 38698223

doi: 10.1007/s00266-024-04047-5

title: Medical Specialist Care Utilization Prior to the Explantation of Cosmetic Silicone Breast Implants: A Nationwide Retrospective Data Linkage Study.

authors

- Lieffering AS

- Ramerman L
- Verheij RA
- Rakhorst HA
- Mureau MAM
- van der Hulst RRWJ
- Hommes JE

journal: Aesthetic plastic surgery

journalAbbr: Aesthetic Plast Surg

year: 2024

pubdate: 2024 Nov

volume: 48

issue: 21

pages: 4404-4413

abstract: BACKGROUND: Explantation is the proposed treatment for breast implant illness (BII). Little is known about which medical specialists are visited and what diagnoses are made before explantation is provided as the treatment. OBJECTIVES: This study investigated medical specialist care utilization in women with cosmetic breast implants who underwent explantation compared to women who chose breast implant replacement surgery and to women without breast implants. METHODS: Retrospective cohort study using data linkage with the Dutch Breast Implant Registry and the Dutch health insurance claims database. Visits to medical specialists were examined over the 3 years before explantation. A total of 832 explantation patients were matched and compared to 1463 breast implant replacement patients and 1664 women without breast implants. RESULTS: Explantation patients were more likely to have visited > 5 different medical specialties compared to both replacement patients (12.3% vs. 5.7%; $p < 0.001$) and women without breast implants (12.3% vs. 3.7%; $p < 0.001$). Among explantation patients, women who underwent explantation because of BII were more likely to have visited > 5 different medical specialties compared to women who underwent explantation because of other reasons (25.0% vs. 11.0%; $p < 0.001$). CONCLUSIONS: Women who underwent explantation of breast implants had higher utilization of medical specialist care in the years before explantation compared to women who underwent breast implant replacement surgery and women without breast implants. Medical specialist care use was especially high among women for whom BII was the registered reason for explantation. These findings suggest further research is needed into the link between BII and the use of medical specialist care. LEVEL OF EVIDENCE III: This journal requires that authors assign a level of evidence to each article. For a full description of these Evidence-Based Medicine ratings, please refer to the Table of Contents or the online Instructions to Authors www.springer.com/00266 .

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11588962/

Record 387

pmcid: PMC10828743

pmid: 38298289

doi: 10.7759/cureus.51444

title: Metastatic Breast Cancer Presenting As Neck Pain in a Pregnant Patient.

authors

- Chu EC
- Yun SMH

journal: Cureus

journalAbbr: Cureus

year: 2024

pubdate: 2024 Jan

volume: 16

issue: 1

pages: e51444

abstract: Breast cancer metastasizing to the spine during pregnancy is a rare occurrence. A 36-year-old woman experienced persistent neck pain during the third trimester of pregnancy. The pain continued until the postpartum period, severely affecting quality of life (QOL). Physical examination revealed a restricted cervical range of motion. Spinal lesions were detected on magnetic resonance imaging. Metastatic breast cancer was confirmed through a biopsy. The patient underwent radiation therapy for spinal metastasis, chemotherapy for breast cancer, and nonsteroidal anti-inflammatory drug therapy for pain. She was referred for chiropractic care and physical rehabilitation. After six weeks of collaborative treatment, the patient experienced significant pain relief and improvement in strength, mobility, and QOL. This case report highlights the need to evaluate unexplained symptoms in pregnant and postpartum women to exclude sinister pathologies. It demonstrates the need for a multidisciplinary approach that combines oncological, chiropractic, and rehabilitative care to optimize the outcomes in patients with complex presentations.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10828743/

Record 388

pmcid: PMC11244267

pmid: 38436980

doi: 10.1210/endrev/bnae003

title: Mini-Puberty, Physiological and Disordered: Consequences, and Potential for Therapeutic Replacement.

authors

- Rohayem J
- Alexander EC
- Heger S
- Nordenström A
- Howard SR

journal: Endocrine reviews

journalAbbr: Endocr Rev

year: 2024

pubdate: 2024 Jul 12

volume: 45

issue: 4

pages: 460-492

abstract: There are 3 physiological waves of central hypothalamic-pituitary-gonadal (HPG) axis activity over the lifetime. The first occurs during fetal life, the second-termed "mini-puberty"-in the first months after birth, and the third at puberty. After adolescence, the axis remains active all through adulthood. Congenital hypogonadotropic hypogonadism (CHH) is a rare genetic disorder characterized by a deficiency in hypothalamic gonadotropin-releasing hormone (GnRH) secretion or action. In cases of severe CHH, all 3 waves of GnRH pulsatility are absent. The absence of fetal HPG axis activation manifests in around 50% of male newborns with micropenis and/or undescended testes (cryptorchidism). In these boys, the lack of the mini-puberty phase accentuates testicular immaturity. This is characterized by a low number of Sertoli cells, which are important for future reproductive capacity. Thus, absent mini-puberty will have detrimental effects on later fertility in these males. The diagnosis of CHH is often missed in infants, and even if recognized, there is no consensus on optimal therapeutic management. Here we review physiological mini-puberty and consequences of central HPG axis

disorders; provide a diagnostic approach to allow for early identification of these conditions; and review current treatment options for replacement of mini-puberty in male infants with CHH. There is evidence from small case series that replacement with gonadotropins to mimic "mini-puberty" in males could have beneficial outcomes not only regarding testis descent, but also normalization of testis and penile sizes. Moreover, such therapeutic replacement regimens in disordered mini-puberty could address both reproductive and nonreproductive implications.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11244267/

Record 389

pmcid: PMC11026943

pmid: 38646367

doi: 10.7759/cureus.56515

title: Mucinous Carcinoma of the Breast: A Case Report.

authors

- Rey J
- Shukla S
- Acharya S
- Gadkari P
- Sihag S

journal: Cureus

journalAbbr: Cureus

year: 2024

pubdate: 2024 Mar

volume: 16

issue: 3

pages: e56515

abstract: This case report presents the diagnostic journey of a 65-year-old female presenting with symptoms suggestive of breast pathology, ultimately diagnosed with mucinous carcinoma, following comprehensive clinical evaluation and histopathological confirmation. Initial assessments indicated a fibroadenoma; however, subsequent histopathological examination revealed mucinous carcinoma, highlighting the importance of histopathological confirmation in establishing definitive diagnoses. The case underscores the challenges in distinguishing between benign and malignant breast lesions based on clinical presentation and imaging findings alone. The multidisciplinary approach facilitated discussions regarding treatment options tailored to the patient's clinical and pathological characteristics. This case emphasizes the significance of a comprehensive diagnostic approach, integrating clinical evaluation, imaging studies, and histopathological analysis, in ensuring accurate diagnosis and guiding optimal management strategies for patients with breast cancer.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11026943/

Record 390

pmcid: PMC10953835

pmid: 38507271

doi: 10.1002/cnr2.2052

title: New insights on testicular cancer prevalence with novel diagnostic biomarkers and therapeutic approaches.

authors

- Sadek KM

- AbdEllatief HY
- Mahmoud SFE
- Alexiou A
- Papadakis M
- Al-Hajeili M
- Saad HM
- Batiha GE

journal: Cancer reports (Hoboken, N.J.)

journalAbbr: Cancer Rep (Hoboken)

year: 2024

pubdate: 2024 Mar

volume: 7

issue: 3

pages: e2052

abstract: BACKGROUND: Testicular cancer (TC), comprising merely 1% of male neoplasms, holds the distinction of being the most commonly encountered neoplasm among young males. RECENT FINDINGS: Most cases of testicular neoplasms can be classified into two main groups, namely germ cell tumors representing approximately 95% of the cases, and sex cord-stromal tumors accounting for about 5% of the cases. Moreover, its prevalence is on the rise across the globe. TC is a neoplastic condition characterized by a favorable prognosis. The advent of cisplatin-based chemotherapeutic agents in the latter part of the 1970s has led to a significant enhancement in the 5-year survival rate, which presently surpasses 95%. Given that TC is commonly detected before reaching the age of 40, it can be anticipated that these individuals will enjoy an additional 40-50 years of life following successful treatment. The potential causes of TC are multifactorial and related to different pathologies. Accurate identification is imperative to guarantee the utmost efficacious and suitable therapy. To a certain degree, this can be accomplished through the utilization of blood examinations for neoplastic indicators; nonetheless, an unequivocal diagnosis necessitates an evaluation of the histological composition of a specimen via a pathologist. CONCLUSION: TC is multifactorial and has various pathologies, therefore this review aimed to revise the prenatal and postnatal causes as well as novel diagnostic biomarkers and the therapeutic strategies of TC.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10953835/

Record 391

pmcid: PMC11769038

pmid: 39852962

doi: 10.3390/toxins17010009

title: Occurrence and Exposure Assessment of Zearalenone in the Zhejiang Province, China.

authors

- Lu Z
- Zhang R
- Wu P
- Zhao D
- Chen J
- Pan X
- Wang J
- Zhang H

- Qi X
- Weng Q
- Ye S
- Zhou B

journal: Toxins

journalAbbr: Toxins (Basel)

year: 2024

pubdate: 2024 Dec 27

volume: 17

issue: 1

pages:

abstract: This study aims to examine the hazards of zearalenone (ZEN) to humans and assess the risk of dietary exposure to ZEN, particularly in relation to precocious puberty in children from the Zhejiang Province. The test results from five types of food from the Zhejiang Province show that corn oil has the highest detection rate of 87.82%. The levels of ZEN do not exceed the existing safety standards in any sample investigated in this study. According to the data from the Food Consumption Survey of Zhejiang Province residents, rice is the primary source of ZEN exposure, accounting for 55.85% of total exposure among all age groups. Based on the 50th exposure percentile, it would take 6.25 years of rice consumption to reach 1 year of safe ZEN exposure. Overall, the majority of the residents in the Zhejiang Province have a low risk of exposure to ZEN. In an extreme case (based on the 95th exposure percentile), the total ZEN exposure from the studied foods with respect to children aged ≤6 years and 7-12 years is 0.38 μg/kg b.w. and 0.26 μg/kg b.w., respectively-both exceeding the safety limit of 0.25 μg/kg b.w. set by the European Food Safety Authority, indicating a potential risk of exposure. Precocious puberty assessments show that ZEN exposure levels in children in the Zhejiang Province are significantly lower than those associated with precocious puberty; thus, precocious puberty is unlikely to occur in this area. Given ZEN's estrogenic effect, it is necessary to monitor the level of ZEN in different food items, revise the relevant standards as needed, and focus on exposure to ZEN in younger age groups.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11769038/

Record 392

pmcid: PMC11268332

pmid: 38959615

doi: 10.1016/j.ijscr.2024.109954

title: One of the biggest and heaviest juvenile bilateral breast fibroadenoma: A case report.

authors

- Al Hakkak SMM
- Al Faham FSM
- Awwady AN
- Al Hakkak MS
- Al Wadees AA

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2024

pubdate: 2024 Aug

volume: 121

issue:

pages: 109954

abstract: INTRODUCTION: Giant juvenile fibroadenomas occurring at a mean age of 13 may be multiple and bilateral, accounting for approximately 0.5 % of all fibroadenomas. The pathogenesis of these tumors is closely linked to hormonal changes during puberty, characterized by increased estrogen stimulation, heightened estrogen receptor sensitivity, and reduced estrogen antagonists. These hormonal factors are pivotal in the rapid growth and substantial size observed in giant juvenile fibroadenomas. CASE PRESENTATION: An adolescent girl presented at the outpatient clinic with significant bilateral breast enlargement, causing redness and discomfort when sitting, leading to difficulty wearing age-appropriate clothing and chest wall pressure. Despite previous consultations attributing the condition to genetic causes, further investigation via radiological ultrasound indicated a probable diagnosis of bilateral breast mass fibroadenoma, occupying a substantial portion of the breast tissue. Consequently, the decision was made to perform bilateral breast surgery to remove the giant masses for histopathological analysis. DISCUSSION: The delayed diagnosis accentuated the case's complexity, highlighting the challenges in effectively identifying and managing giant fibroadenomas in adolescents. Despite the initial absence of alarming symptoms, these fibroadenomas' sheer size and impact underscored the importance of early detection and comprehensive evaluation in similar clinical presentations. CONCLUSION: The surgeon must emphasize meticulous planning when deciding on the surgical approach for removing a giant juvenile fibroadenoma. This planning is crucial for preserving breast functionality, achieving a satisfactory cosmetic outcome, and addressing the psychological distress of young patients. Early detection and excision are imperative to safeguard breast tissue.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11268332/

Record 393

pmcid: PMC11739267

pmid: 39317861

doi: 10.1007/s00266-024-04404-4

title: Optimizing Gynecomastia Correction Surgery: Efficacy and Safety of Tumescent Local Anesthesia Approach.

authors

- Tettamanzi M
- Filigheddu E
- Ziani F
- Arrica G
- Trignano C
- Rubino C
- Trignano E

journal: Aesthetic plastic surgery

journalAbbr: Aesthetic Plast Surg

year: 2024

pubdate: 2024 Dec

volume: 48

issue: 23

pages: 5067-5073

abstract: BACKGROUND: Tumescent local anesthesia (TLA) involves infusing a saline solution containing lidocaine and epinephrine into tissues to achieve localized anesthesia and vasoconstriction. While liposuction under general anesthesia remains the most used treatment, we introduce a novel TLA approach for

gynecomastia surgery, drawing from our extensive experience in recent years. METHODS: Between the years 2010 and 2023, we performed gynecomastia surgery on 60 male patients under TLA. The gynecomastia was treated by liposuction plus periareolar excision technique. Liposuction was carried out on both breasts in every case, regardless of whether the gynecomastia was bilateral or unilateral. The tumescent solution consisted of 25 mL of 2% lidocaine, 8 mEq of sodium bicarbonate, and 1 mL of epinephrine (1 mg/1 mL) in 1000 mL of 0.9% saline solution. The solution was infiltrated between the pectoral fascia and the mammary gland, and then the surgery was carried out. RESULTS: The average volume of tumescent solution infiltrated during TLA was 300 mL per breast. There were no reports of adrenaline or lidocaine toxicity, and no cases required a conversion to general anesthesia. Patients experienced no pain or discomfort during the preoperative infiltration or surgical procedure. We observed a major postoperative complications rate of 6.7%, represented by three incident of hematoma and one case of seroma. A minor complication rate of 5% was observed: two cases of retraction of the NAC and one case of gynecomastia recurrence, the latter undergoing an additional combination procedure with liposuction and subcutaneous mastectomy. Follow-up time ranged from 30 days to 1 year. CONCLUSIONS: We developed a new outpatient surgical method for gynecomastia using liposuction and periareolar excision under tumescent local anesthesia. This technique, supported by a comprehensive rehabilitation plan, proved a successful and quick recovery, and high patient satisfaction. Our results suggest it is a feasible and effective option, warranting further consideration in gynecomastia treatment strategies. LEVEL OF EVIDENCE IV: This journal requires that authors assign a level of evidence to each article. For a full description of these Evidence-Based Medicine ratings, please refer to the Table of Contents or the online Instructions to Authors www.springer.com/00266.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11739267/

Record 394

pmcid: PMC11619261

pmid: 39633291

doi: 10.1186/s12887-024-05273-1

title: Outcomes of neuroendoscopy for suprasellar arachnoid cysts: a single center experiences.

authors

- Kuzucu P
- Kale A
- Türkmen T
- Görür E
- Börcek AÖ

journal: BMC pediatrics

journalAbbr: BMC Pediatr

year: 2024

pubdate: 2024 Dec 5

volume: 24

issue: 1

pages: 797

abstract: BACKGROUND: Arachnoid cysts are extra parenchymal, intra-arachnoid fluid collections of unknown origin, similar in content to cerebrospinal fluid. Suprasellar arachnoid cysts are a rarer localization resulting from abnormalities of the Liliequist membrane or cystic dilatation of the interpeduncular cisterna. We aimed to contribute to the literature by presenting a series of suprasellar arachnoid cyst cases with endoscopic intervention and long-term results. METHODS: This study analyzed radiographic data, clinical symptoms, surgical records, and videos of giant arachnoid cysts that underwent endoscopic surgery between 2013 and 2023. Cyst volumes and Evan's indexes were calculated both preoperatively and postoperatively. RESULTS: The patient population consisted of 3 males (21.4%, 3/14 patients) and 11 females (78.5%, 11/14 patients).

Their age at the time of surgery ranged from 1 year to 56 years (mean: 14.4 years). Of the total 14 patients, 4 (28.5%, 4/14 patients) were adults (age ≥ 18 years), and 10 (71.4%, 10/14 patients) were pediatric (age < 18 years). The most common presenting complaints were gait disturbance (21.4%, 3/14 patients), headache (21.4%, 3/14 patients), increased head circumference (14.2%, 2/14 patients), hormonal disorders (14.2%, 2/14 patients), seizures (14.2%, 2/14 patients), and forgetfulness (7.1%, 1/14 patients). Ventriculocystocisternostomy was performed in all patients. The most common perioperative complication was bleeding controlled by irrigation (28.5%, 4/14 patients). The mean follow-up period was 63.8 months (min:12 months-max:125 months). In the postoperative long-term follow-up, the preoperative neurologic examinations of the patients showed complete recovery, and hormonal problems returned to normal. CONCLUSIONS: Endoscopic options for the surgery of rare giant suprasellar arachnoid cysts, especially ventriculocisternostomy, are safe, less invasive, and successful, with fewer recurrences and the need for a second operation in the long-term results of patients who underwent ventriculocisternostomy according to our experience.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11619261/

Record 395

pmcid: PMC10949056

pmid: 38510229

doi: 10.31486/toj.23.0079

title: Outcomes of Reduction Mammaplasty in Adolescents vs Average-Age Patients: A 3-Year Single-Center Retrospective Analysis.

authors

- Hajebian HH
- Puyana S
- Babycos CR
- Friel MT

journal: Ochsner journal

journalAbbr: Ochsner J

year: 2024

pubdate: 2024 Spring

volume: 24

issue: 1

pages: 14-21

abstract: Background: The onset of macromastia symptomatology occurs most often at puberty, yet most females undergo breast reduction surgery during the fifth decade of life. Adolescent patients with macromastia may benefit from reduction mammaplasty, yet outcome data are limited to a small number of institutions. Methods: We conducted a retrospective medical records review of all patients who underwent reduction mammaplasty at our institution during the years 2016 to 2019. Patients were divided into 2 cohorts based on age: adolescent (10 to 24 years) and average age (≥44 years). Demographics and outcome measures were collected from follow-up evaluations at 1-week, 1-month, 3-month, 6-month, and 12-month intervals postoperatively. Results: A total of 141 patients met the inclusion criteria for the study. Mean age at surgery was 19 ± 3.2 years in the adolescent group and 53 ± 7.4 years in the average-age group. No significant differences in complications related to wound healing (42.9% vs 50.0%, P =0.418) or total postoperative complications (18.4% vs 19.6%, P =0.863) were found between adolescent and average-age patients, respectively. Conclusion: Complications related to wound healing are common in reduction mammaplasty, although rates of life-threatening complications are rare. In this 3-year review comparing the outcomes of adolescent vs average-age patients who underwent reduction mammaplasty at the same institution, no significant differences in postoperative complication rates were found. Our data suggest that adolescent patients with macromastia should not defer reduction mammaplasty out of concern for higher complication rates because of age alone.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10949056/

Record 396

pmcid: PMC11260271

pmid: 39035786

doi: 10.1093/jbmrpl/ziae083

title: Parathyroid hormone-related peptide induced hypercalcemia of pregnancy due to mammary hyperplasia.

authors

- Jodeh W
- Sparks PJ
- Higgins JM
- Tom A
- Anilovich N
- Moit H
- Korff L
- Hadad I
- Wang X
- Imel EA
- Donegan DM

journal: JBMR plus

journalAbbr: JBMR Plus

year: 2024

pubdate: 2024 Aug

volume: 8

issue: 8

pages: ziae083

abstract: Maternal Parathyroid Hormone-related Protein (PTHrP) is involved in the placental transport of calcium. Autonomous overproduction of PTHrP is a rare cause of hypercalcemia in pregnancy. Prior cases of PTHrP-induced hypercalcemia in pregnancy have been managed with either dopamine agonists, fetal delivery, termination of pregnancy, or mastectomy. However, PTHrP level normalization following mastectomy has not previously been documented. Herein, we present a 39-year-old female hospitalized at 19 weeks of gestation for acute encephalopathy due to PTHrP induced hypercalcemic crisis (calcium 15.8 mg/dL, PTHrp 46.5 pmol/L [normal 0-3.4]). Mammary hyperplasia resulting in gigantomastia significantly impaired her ability to ambulate and perform activities of daily living. She remained hypercalcemic during hospitalization despite aggressive hydration, calcitonin, and 2 weeks of dopamine agonist treatment. Bisphosphonate therapy was not administered due to pregnancy and potential effects on the fetus. Our patient underwent bilateral mastectomy along with excision of a large axillary mass. The pathology of all three specimens revealed mammary stromal hyperplasia. PTHrP was undetectable on post-op day 2 and calcium normalized by post-op day 3. At discharge, she was able to ambulate independently. To our knowledge, this is the first reported case of PTHrP induced hypercalcemia related to gigantomastia, documenting resolution of hypercalcemia, and PTHrP levels following mastectomy. Mastectomy is a potential option in the second trimester for pregnant patients with PTHrP induced severe hypercalcemia due to gigantomastia, refractory to treatment with dopamine agonist therapy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11260271/

Record 397

pmcid: PMC11326481

pmid: 38473389

doi: 10.3390/cancers16051032

title: Peptide Therapeutics: Unveiling the Potential against Cancer-A Journey through 1989.

authors

- Al Musaimi O

journal: Cancers

journalAbbr: Cancers (Basel)

year: 2024

pubdate: 2024 Mar 2

volume: 16

issue: 5

pages:

abstract: The United States Food and Drug Administration (FDA) has approved a plethora of peptide-based drugs as effective drugs in cancer therapy. Peptides possess high specificity, permeability, target engagement, and a tolerable safety profile. They exhibit selective binding with cell surface receptors and proteins, functioning as agonists or antagonists. They also serve as imaging agents for diagnostic applications or can serve a dual-purpose as both diagnostic and therapeutic (theragnostic) agents. Therefore, they have been exploited in various forms, including linkers, peptide conjugates, and payloads. In this review, the FDA-approved prostate-specific membrane antigen (PSMA) peptide antagonists, peptide receptor radionuclide therapy (PRRT), somatostatin analogs, antibody-drug conjugates (ADCs), gonadotropin-releasing hormone (GnRH) analogs, and other peptide-based anticancer drugs are analyzed in terms of their chemical structures and properties, therapeutic targets and mechanisms of action, development journey, administration routes, and side effects.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11326481/

Record 398

pmcid: PMC11040274

pmid: 38660487

doi: 10.1210/jcemcr/luae050

title: Persistent Gynecomastia due to Short-term Low-dose Finasteride for Androgenetic Alopecia.

authors

- Farkas HS
- Jee YH
- Szymczuk V
- Leschek EW

journal: JCEM case reports

journalAbbr: JCEM Case Rep

year: 2024

pubdate: 2024 May

volume: 2

issue: 5

pages: luae050

abstract: We report a case of persistent gynecomastia in a healthy 20-year-old man after 1 month of low-dose finasteride. Finasteride was discontinued after 2 months, and gynecomastia was unchanged 5 months after drug

withdrawal. The gynecomastia regressed but did not resolve after 6 months of treatment with raloxifene, a selective estrogen receptor modulator. One year later, bilateral mammoplasty was performed to remove the remaining breast tissue. Finasteride, a 5-alpha-reductase inhibitor, is widely used for the treatment of androgenetic alopecia. Gynecomastia is an expected side effect of this therapy given its mechanism of action. However, only 8 cases of gynecomastia have been reported with low-dose (1 mg daily) finasteride treatment since its approval for androgenetic alopecia in 1997. This raises the concern that gynecomastia resulting from low-dose finasteride is significantly underreported, causing inadequately informed patients. Further, because of the risk of gynecomastia, it is important for prescribing physicians to counsel patients regarding this complication and to consider early intervention when finasteride-induced gynecomastia first arises to prevent fibrosis and thus irreversible gynecomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11040274/

Record 399

pmcid: PMC10885350

pmid: 38405141

doi: 10.3389/fendo.2024.1323947

title: Pineal cysts may promote pubertal development in girls with central precocious puberty: a single-center study from China.

authors

- Yuan S
- Lin Y
- Zhao Y
- Du M
- Dong S
- Chen Y
- Wei H

journal: Frontiers in endocrinology

journalAbbr: Front Endocrinol (Lausanne)

year: 2024

pubdate: 2024

volume: 15

issue:

pages: 1323947

abstract: INTRODUCTION: Pineal cysts have long been considered a benign intracranial variation. However, in our clinical practice, it has been observed that some children with central precocious puberty (CPP) who have pineal cysts experience rapid progression in adolescent development. In recent years, there has been a significant increase in the prevalence of CPP in girls, leading to more diagnoses of CPP among children with pineal cysts. Despite this, there is no consensus regarding whether pineal cysts contribute to CPP as one of its organic factors. This study aimed to analyze the clinical characteristics of pineal cysts in children with CPP and explore the potential effects of pineal cysts on puberty development. METHODS: This single-center study retrospectively analyzed clinical data from girls aged 3 to 10 years who underwent head/pituitary magnetic resonance imaging at the Children's Hospital Affiliated to Zhengzhou University between 2019 and 2022. The study categorized the detection rates of pineal cysts based on systematic disease classification and compared the rates of cyst detection between girls diagnosed with CPP and those without CPP. Subsequently, CPP-diagnosed girls with pineal cysts were examined. Among CPP-diagnosed girls meeting the study's criteria, those with pineal cysts formed the 'cyst group,' while those without cysts were matched in a 1:1 ratio based on age and body mass index to form the 'non-cyst group.' Comparative analyses were conducted to assess the clinical

characteristics between these two groups. CPP-diagnosed girls with cysts were further subdivided into three groups according to cyst size (≤5 mm, 5.1-9.9 mm, and ≥10 mm) to investigate potential differences in clinical characteristics among these subgroups. The study involved an analysis of clinical data from girls diagnosed with CPP and included imaging follow-ups to explore the progression of pineal cysts over time. RESULTS: Among the 23,245 girls who underwent head/pituitary magnetic resonance imaging scans, the detection rate of pineal cysts was 3.6% (837/23,245), with most cases being associated with endocrine diseases. The detection rate of pineal cysts in CPP patients was 6.4% (262/4099), which was significantly higher than the 3.0% (575/19,146) in patients without CPP. In comparison to the non-cyst group, the cyst group exhibited statistically significant increases in estradiol levels, peak luteinizing hormone (LH) levels, peak LH/follicle-stimulating hormone (FSH) ratios, uterine body length, and cervix length ($P < 0.001$). As cyst size increased, there were significant rises in LH peak, peak LH/FSH ratio, uterine body length, and cervical length ($P < 0.01$). Estradiol levels and left ovarian volume also showed an increasing trend ($P < 0.05$). Among girls who underwent follow-up imaging, 26.3% (5/19) exhibited an increase in cyst size. CONCLUSION: Pineal cysts are relatively common in children with CPP. They may affect the pubertal development process, with larger cysts correlating to faster pubertal development. Therefore, the authors hypothesize that pineal cysts may trigger CPP in some cases, especially when the cysts are larger than 5 mm in size, as indicated by our data.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10885350/

Record 400

pmcid: PMC11535441

pmid: 39502127

doi: 10.1002/ccr3.9541

title: Pituitary hyperplasia secondary to primary hypothyroidism in adolescents: A medical case report and literature review.

authors

- Yuan X
- Zhu J
- Su X
- Tan H
- Wang S
- Zheng X
- Ding Y
- Li S

journal: Clinical case reports

journalAbbr: Clin Case Rep

year: 2024

pubdate: 2024 Nov

volume: 12

issue: 11

pages: e9541

abstract: Prompt and precise diagnosis of pituitary hyperplasia secondary to primary hypothyroidism (PHPH) is crucial to avoid unwarranted pituitary surgery and potential permanent impairment. Although PHPH may present similarly to pituitary adenoma, it is responsive to thyroxine therapy, underscoring the critical role of differential diagnosis in the treatment of adolescent patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11535441/

Record 401

pmcid: PMC11436444

pmid: 39331227

doi: 10.1007/s40519-024-01688-y

title: Position statement of Italian Society of Obesity (SIO): Gestational Obesity.

authors

- Barrea L
- Camastra S
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journal: Eating and weight disorders : EWD

journalAbbr: Eat Weight Disord

year: 2024

pubdate: 2024 Sep 27

volume: 29

issue: 1

pages: 61

abstract: PURPOSE: Gestational obesity (GO) presents a multifaceted challenge to maternal and fetal health, with an escalating prevalence and far-reaching consequences extending beyond pregnancy. This perspective statement by the Italian Society of Obesity (SIO) provides current insights into the diagnosis, maternal and fetal impacts, and treatment strategies for managing this pressing condition. METHODS: This article provides a comprehensive review of the maternal and fetal effects of GO and provides suggestions on strategies for management. Comprehensive review was carried out using the MEDLINE/PubMed, CINAHL, EMBASE, and Cochrane Library databases. RESULTS: The diagnosis of GO primarily relies on pre-pregnancy body mass index (BMI), although standardized criteria remain contentious. Anthropometric measures and body composition assessments offer valuable insights into the metabolic implications of GO. Women with GO are predisposed to several health complications, which are attributed to mechanisms such as inflammation and insulin resistance. Offspring of women with GO face heightened risks of perinatal complications and long-term metabolic disorders, indicating intergenerational transmission of obesity-related effects. While nutritional interventions are a cornerstone of management, their efficacy in mitigating complications warrants further investigation. Additionally, while pharmacological interventions have been explored in other contexts, evidence on their safety and efficacy specifically for GO remains lacking, necessitating further investigation. CONCLUSION: GO significantly impacts maternal and fetal health, contributing to both immediate and long-term complications. Effective management requires a multifaceted approach, including precise diagnostic criteria, personalized nutritional interventions, and potential pharmacological treatments. These findings underscore the need for individualized care strategies and further research to optimize outcomes for mothers and their offspring are needed. Enhanced understanding and management of GO can help mitigate its intergenerational effects, improving public health outcomes. LEVEL OF EVIDENCE: Level V narrative review.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11436444/

Record 402

pmcid: PMC11097837

pmid: 38749697

doi: 10.1136/bmjopen-2023-073951

title: Practice of breast self-examination and associated factors among women of reproductive age in the North Shoa Zone, Oromia, Ethiopia, 2022: a convergent mixed-methods study.

authors

- Hussein D
- Oyato BT
- Gashaw K
- Geleta TA
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journal: BMJ open

journalAbbr: BMJ Open

year: 2024

pubdate: 2024 May 15

volume: 14

issue: 5

pages: e073951

abstract: OBJECTIVE: To assess practice of breast self-examination (BSE) and associated factors among women of reproductive age in the North Shoa Zone, Oromia, Ethiopia. DESIGN: Community-based cross-sectional study with a convergent mixed-methods approach. SETTING: The study was conducted in the North Shoa Zone of Oromia, Central Ethiopia, between 1 May 2022 and 30 June 2022. PARTICIPANTS: 1076 women of reproductive age were selected using simple random sampling technique for the quantitative study. For the qualitative part, a total of 46 women were approached purposively to elicit rich ideas and insight into the issue. The quantitative data were collected using an interviewer-administered questionnaire, and focus group discussions were used for the qualitative part. Quantitative data were analysed using SPSS V.26. An adjusted OR with a 95% CI was employed. Thematic analysis approach was applied for the qualitative data analysis. OUTCOME MEASURES: Participants were interviewed to respond whether they practised BSE. RESULTS: Overall, 192 (18.2%; 15.7%-20.5%) of the participants had ever performed BSE. Having a family history of breast cancer (BC) (adjusted OR (AOR)=6.9, 95% CI 4.6 to 10.3), being knowledgeable on BSE (AOR=3, 95% CI 1.9 to 4.3), having high perceived susceptibility (AOR=1.7, 95% CI 1.2 to 2.5), having high self-efficacy (AOR=1.5, 95% CI 1.1 to 2.3) and having a high perceived benefit to BSE (AOR=1.5, 95% CI 1.1 to 2.3) were significantly associated with increased odds of BSE practice.Four main themes emerged from the qualitative analysis: BC and BSE-related knowledge, perceived benefits of treatment, barriers to BSE practice and enablers of BSE practice. CONCLUSION: The practice of BSE in this area was considerably low. The North Shoa Zonal Health Office and other stakeholders should disseminate instructional materials that cover the techniques and benefits of regularly performing BSE. Healthcare professionals should engage in the community to address the obstacles women face in practising BSE.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11097837/

Record 403

pmcid: PMC10850763

pmid: 38333642

doi: 10.2147/PPA.S440689

title: Preferences for Tailored Support - Patients' and Health Care Professionals' Experiences Regarding Symptoms and Self-Management Strategies During the First Year After Curatively Intended Prostate Cancer Treatment.

authors

- Kelmendi N
- Nilsson M
- Taloyan M
- Sundberg K
- Langius-Eklöf A
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journal: Patient preference and adherence

journalAbbr: Patient Prefer Adherence

year: 2024

pubdate: 2024

volume: 18

issue:

pages: 275-288

abstract: PURPOSE: There is an increase in the number of men undergoing screening for prostate cancer, and advancements in treatments, which implies current knowledge about symptoms and self-management. This study aims to explore experiences of symptom distress, and self-management strategies during the first year after curatively intended treatment for prostate cancer, as identified by patients and health care professionals. METHODS: A qualitative design was used, including data triangulation from individual interviews with patients (n =17) and one focus group interview with healthcare professionals (n =5). Thematic analysis was used. RESULTS: The two main themes were identified: living with the consequences of treatment and navigating a new situation. Living with the consequences of treatment illustrated how losing control of bodily functions such as bladder, bowel, and sexual functions interfered with daily life. A stigma around the disease was described, and a life living in an unfamiliar body challenged ideas of masculinity. The first months after treatment ended was a distressing period related to the abruption in frequent contact with healthcare providers, and concerns about the future. The second theme, navigating a new situation , illustrates that self-management strategies varied, due to individual factors as did the need for tailored information and support provided from healthcare professionals and family, which was highly valued. Information and support were described as complex topics and healthcare professionals emphasized the need for appropriate education for staff to provide proper support to men after ended treatment. CONCLUSION: Lingering symptoms and concerns were evident during the first year after treatment. Self-management strategies varied, and timely and tailored information and support during the first year were considered highly valued, important, and preferred by patients. Our results indicate that support should be offered immediately after curatively intended treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10850763/

Record 404

pmcid: PMC11237254

pmid: 38286512

doi: 10.15441/ceem.23.089

title: Preoxygenation and apneic oxygenation in emergency airway management.

authors

- Barbosa A
- Mosier JM

journal: Clinical and experimental emergency medicine

journalAbbr: Clin Exp Emerg Med

year: 2024

pubdate: 2024 Jun

volume: 11

issue: 2

pages: 136-144

abstract: Preoxygenation during the peri-intubation period is now considered a critical aspect of rapid sequence intubation and an important skill for emergency medicine and critical care providers. Peri-intubation hypoxemia carries significant risk, including cardiac arrest, and care must be taken for appropriate management including through apnea and initiation of laryngoscopy. Appropriate selection of preoxygenation devices should depend on underlying physiology to optimize oxygenation prior to intubation attempts. A PubMed MEDLINE search was completed with selection of articles from March 2008 to March 2023 describing various techniques for preoxygenation for intubation in the critical care and operating room setting with pregnant and obese patient populations included. Prehospital and pediatric populations were excluded in this review. This review provides an overview of methods of preoxygenation with their clinical indications as well as methods for determining end points to preoxygenation and apneic oxygenation. An overview of approaches to preoxygenation was included for patients considered to have a physiologically difficult airway and obese and pregnant patient populations.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11237254/

Record 405

pmcid: PMC11520314

pmid: 39473792

doi: 10.1039/d4ra04866k

title: Prevalence and health risk evaluations of mycotoxins in drinking water sources in Nigeria.

authors

- Koko DT
- Alfred MO
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- Okonofua F
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journal: RSC advances

journalAbbr: RSC Adv

year: 2024

pubdate: 2024 Oct 23

volume: 14

issue: 46

pages: 34435-34447

abstract: Mycotoxins in drinking water are neglected pollutants that have serious health implications when ingested. Common mycotoxins with health concerns include deoxynivalenol (DON), ochratoxin A (OTA), and zearalenone (ZEN). This study considers the distribution and apparent health risks of these mycotoxins in drinking water sources (groundwater, surface water, bottled water, sachet water) in three Southwest Nigeria States: Osun, Oyo, and Lagos States, using the UHPLC-ESI-QTOF. ZEN and DON were found in all 95 water samples across all three states. ZEN in sachet water samples has the highest mean concentrations, with those from Osun, Oyo, and Lagos States having concentrations of 14.96 ± 4.46, 8.59 ± 3.86, and 10.56 ± 2.84 μg L -1 , respectively. In contrast, the mean concentrations of all three mycotoxins (± Standard Error of Mean) in bottled water samples are the lowest across the three States. The mean concentrations of OTA in sachet water samples (2.93 ± 0.79, 1.24 ± 0.40, and 3.01 ± 1.50 μg L -1) are slightly higher than those in bottled water (1.47 ± 0.28, 1.53 ± 0.31, and 0.75 ± 0.31 μg L -1) for Osun, Oyo, and Lagos States, respectively. Groundwater samples across the three States had the lowest average pH values, below the WHO's lowest limit of 6.5. Principal Component Analysis studies indicate that all three mycotoxins in water samples are closely associated by source. Possible human exposure values for ZEN suggest some health concerns, especially with the use of sachet water type based on the estrogenicity of this mycotoxin. However, OTA values for all water samples analyzed, are much lower than the tolerable daily intake (TDI) of 4.73 μg kg -1 bw per day provided by European Food Safety Auhtority (EFSA). The levels of DON in all water sample types were of no serious health concern. However, human exposure levels to all three mycotoxins in bottled water fall within safe limits of health-based guidance values (HBGV) indicating that this drinking water type is better treated, unlike sachet water. Nevertheless, stakeholders need to re-examine water quality with respect to these mycotoxins and adopt stringent guidelines and new water treatment strategies to provide consumers with safe drinking water in line with the UN SDG #6.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11520314/

Record 406

pmcid: PMC10932624

pmid: 38476640

doi: 10.1155/2024/9679935

title: Prevalence of Hypogonadism and Associated Risk Factors among Newly Diagnosed ART Naïve HIV-Infected Males in Mwanza, Tanzania.

authors

- Iddi S
- Dika H
- Kidenya B
- Kalluvya S

journal: International journal of endocrinology

journalAbbr: Int J Endocrinol

year: 2024

pubdate: 2024

volume: 2024

issue:

pages: 9679935

abstract: BACKGROUND: Hypogonadism is frequent among HIV-infected males and might have significant clinical impact leading to sexual impairment and metabolic derangement. There is limited information about the magnitude of hypogonadism and its associated factors among people living with HIV in Tanzania. We aimed to determine the prevalence of hypogonadism and associated risk factors among newly diagnosed ART naïve HIV-infected men in Mwanza, Tanzania. METHODS: Newly diagnosed ART naïve HIV-infected men were enrolled at Voluntary Counseling and Testing Centres of four selected hospitals in the Mwanza region and subjected to

thorough clinical and general physical examination including anthropometric measurements. A prestructured questionnaire was used to collect sociodemographic characteristics and clinical data. Serum total testosterone, follicle-stimulating hormone, luteinizing hormone, and estradiol were estimated. Serum total testosterone <300 ng/dl or testosterone >300 ng/dl with high LH and FSH (compensatory hypogonadism) was taken as markers of hypogonadism. Data were analyzed using STATA version 15. RESULTS: Of the 388 enrolled participants, hypogonadism was found in 47.9%, with secondary hypogonadism (83.9%, 156/186) being the most frequent form. Logistic regression analysis showed a significant association between hypogonadism and CD4+ count (OR 2.0; 95% CI 1.1-3.6; p =0.022), decreased libido (OR 1.6; 95% CI 1.1-2.4; p =0.024), age of above 46 years (OR 2.3; 95% CI 1.1-4.6; p =0.023), herbal medicine use (OR 2.4; 95% CI 1.5-3.9; p < 0.001), WHO clinical stage 3 (OR 2.7; 95% CI 1.4-5.2; p =0.003), and weight loss (OR 1.8; 95% CI 1.1-3.0; p =0.016). CONCLUSION: Hypogonadism was found in nearly half (47.9%) of ART naïve HIV-infected men. The majority (83.9%) had secondary hypogonadism. There was a significant association of hypogonadism with older age, herbal medicine use, weight loss, advanced clinical stage, CD4+ count, and decreased libido.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10932624/

Record 407

pmcid: PMC11113035

pmid: 38784244

doi: 10.1177/2050313X241255808

title: Primary angiosarcoma of the breast in a 21-year-old female: A case report.

authors

- Wijetilake B
- Senavirathna J
- Wijesinghe A
- Fernando T
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- Sosai C
- Nandasena M
- Wijesinghe K

journal: SAGE open medical case reports

journalAbbr: SAGE Open Med Case Rep

year: 2024

pubdate: 2024

volume: 12

issue:

pages: 2050313X241255808

abstract: We present a case of primary angiosarcoma, an exceedingly rare malignant breast lesion, in a 21-year-old female patient with unilateral breast enlargement. Primary angiosarcoma is an aggressive form of breast cancer with variable presentation and limited clinical experience due to the rarity of the disease. Despite an initial diagnostic challenge, this report showcases the importance of a systematic diagnostic approach and multidisciplinary management in the context of uncommon breast lesions in young patients. Our patient underwent a mastectomy followed by breast reconstruction, achieving favourable cosmetic outcomes. This case serves as a contribution to our understanding of the diagnostic considerations surrounding angiosarcoma of the breast in a young patient.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11113035/

Record 408

pmcid: PMC11593909

pmid: 39596206

doi: 10.3390/ijms252212139

title: Primary Osteoporosis Induced by Androgen and Estrogen Deficiency: The Molecular and Cellular Perspective on Pathophysiological Mechanisms and Treatments.

authors

- Hsu SH
- Chen LR
- Chen KH

journal: International journal of molecular sciences

journalAbbr: Int J Mol Sci

year: 2024

pubdate: 2024 Nov 12

volume: 25

issue: 22

pages:

abstract: Primary osteoporosis is closely linked to hormone deficiency, which disrupts the balance of bone remodeling. It affects postmenopausal women but also significantly impacts older men. Estrogen can promote the production of osteoprotegerin, a decoy receptor for RANKL, thereby preventing RANKL from activating osteoclasts. Furthermore, estrogen promotes osteoblast survival and function via activation of the Wnt signaling pathway. Likewise, androgens play a critical role in bone metabolism, primarily through their conversion to estrogen in men. Estrogen deficiency accelerates bone resorption through a rise in pro-inflammatory cytokines (IL-1, IL-6, TNF-α) and RANKL, which promote osteoclastogenesis. In the classic genomic pathway, estrogen binds to estrogen receptors in the cytoplasm, forming a complex that migrates to the nucleus and binds to estrogen response elements on DNA, regulating gene transcription. Androgens can be defined as high-affinity ligands for the androgen receptor; their combination can serve as a ligand-inducible transcription factor. Hormone replacement therapy has shown promise but comes with associated risks and side effects. In contrast, the non-genomic pathway involves rapid signaling cascades initiated at the cell membrane, influencing cellular functions without directly altering gene expression. Therefore, the ligand-independent actions and rapid signaling pathways of estrogen and androgen receptors can be harnessed to develop new drugs that provide bone protection without the side effects of traditional hormone therapies. To manage primary osteoporosis, other pharmacological treatments (bisphosphonates, teriparatide, RANKL inhibitors, sclerostin inhibitors, SERMs, and calcitonin salmon) can ameliorate osteoporosis and improve BMD via actions on different pathways. Non-pharmacological treatments include nutritional support and exercise, as well as the dietary intake of antioxidants and natural products. The current study reviews the processes of bone remodeling, hormone actions, hormone receptor status, and therapeutic targets of primary osteoporosis. However, many detailed cellular and molecular mechanisms underlying primary osteoporosis seem complicated and unexplored and warrant further investigation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11593909/

Record 409

pmcid: PMC11466302

pmid: 39391877

doi: 10.3389/fendo.2024.1464803

title: Primary ovarian insufficiency: update on clinical and genetic findings.

authors

- Federici S
- Rossetti R
- Moleri S
- Munari EV
- Frixou M
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- Persani L

journal: Frontiers in endocrinology

journalAbbr: Front Endocrinol (Lausanne)

year: 2024

pubdate: 2024

volume: 15

issue:

pages: 1464803

abstract: Primary ovarian insufficiency (POI) is a disorder of insufficient ovarian follicle function before the age of 40 years with an estimated prevalence of 3.7% worldwide. Its relevance is emerging due to the increasing number of women desiring conception late or beyond the third decade of their lives. POI clinical presentation is extremely heterogeneous with a possible exordium as primary amenorrhea due to ovarian dysgenesis or with a secondary amenorrhea due to different congenital or acquired abnormalities. POI significantly impacts non only on the fertility prospect of the affected women but also on their general, psychological, sexual quality of life, and, furthermore, on their long-term bone, cardiovascular, and cognitive health. In several cases the underlying cause of POI remains unknown and, thus, these forms are still classified as idiopathic. However, we now know the age of menopause is an inheritable trait and POI has a strong genetic background. This is confirmed by the existence of several candidate genes, experimental and natural models. The most common genetic contributors to POI are the X chromosome-linked defects. Moreover, the variable expressivity of POI defect suggests it can be considered as a multifactorial or oligogenic defect. Here, we present an updated review on clinical findings and on the principal X-linked and autosomal genes involved in syndromic and non-syndromic forms of POI. We also provide current information on the management of the premature hypoestrogenic state as well as on fertility preservation in subjects at risk of POI.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11466302/

Record 410

pmcid: PMC11544667

pmid: 39511936

doi: 10.1177/03000605241293684

title: Pseudoangiomatous stromal hyperplasia causing bilateral galactomastia with short-term recurrence after surgery: a case report.

authors

- Lian Z
- Chen X
- Liu C

journal: The Journal of international medical research

journalAbbr: J Int Med Res

year: 2024

pubdate: 2024 Nov

volume: 52

issue: 11

pages: 3000605241293684

abstract: Pseudoangiomatous stromal hyperplasia (PASH) is a rare, benign breast lesion characterized by collagen proliferation, often identified as an incidental microscopic finding. Clinically, it may present as a palpable, well-defined breast mass or, in rare instances, as a diffuse, bilateral process leading to significant breast enlargement. We herein report a case of extensive diffuse PASH with short-term recurrence following lesion resection at another hospital. Because of the severity of symptoms, the patient underwent bilateral mastectomy at our facility. This article reviews the clinical, radiologic, and pathologic features of PASH and emphasizes the importance of an accurate preoperative imaging diagnosis for guiding appropriate treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11544667/

Record 411

pmcid: PMC10798276

pmid: 38250333

doi: 10.1080/23320885.2024.2303993

title: Pseudoangiomatous stromal hyperplasia: a rare cause of gynecomastia in men.

authors

- Saoud S
- Arreyouchi D
- Ankiz A
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- Karich N
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- Oufkir AA

journal: Case reports in plastic surgery & hand surgery

journalAbbr: Case Reports Plast Surg Hand Surg

year: 2024

pubdate: 2024

volume: 11

issue: 1

pages: 2303993

abstract: A 17-year-old male with chest malformation and left breast enlargement underwent surgery for gynecomastia. Histological examination revealed mammary fibrous stroma with ductal hyperplasia and features of pseudoangiomatous stromal hyperplasia. Postoperative follow-up showed no complications, but 8 months later, the patient experienced a mild recurrence with enlargement of the nipple-areolar complex. Although recommended for secondary glandular resection, the patient declined further surgery.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10798276/

Record 412

pmcid: PMC10953823

pmid: 38510076

doi: 10.3389/fped.2024.1367550

title: Pubertal gynecomastia incidence among 530,000 boys: a cross sectional population based study.

authors

- Berger O
- Hornik-Lurie T
- Talisman R

journal: Frontiers in pediatrics

journalAbbr: Front Pediatr

year: 2024

pubdate: 2024

volume: 12

issue:

pages: 1367550

abstract: BACKGROUND: Adolescent gynecomastia, a benign proliferation of male breast tissue, can lead to psychological issues during adolescence. The prevalence varies widely (4%-69%). The incidence peaks are during neonatal, pubertal, and senescent periods. Its affect on emotional well-being necessitates understanding and occasional intervention. This study aimed to determine the incidence of gynecomastia among male adolescents aged 12-15 years. METHODS: A retrospective cross-sectional study utilized the Clalit Health Care Services database (2008-2021) with a population of approximately 4.5 million. Participants aged 12-15 years were included if diagnosed with gynecomastia (International classification of diseases-9 code 611.1) and having a body mass index (BMI) measurement and no obesity diagnosis (ICD9 code 278.0). Data analysis included incidence rates and associations with ethnicity, age, BMI, and socioeconomic status. RESULTS: 531,686 participants included with an incidence of 1.08%. Of all participants, 478,140 had a BMI ≤ 25 with an incidence of 0.7%, and 0.25%-0.35% yearly, and 70% of gynecomastia patients were aged 13-14 years. The prevalence of gynecomastia differed between Jews (1.28%) and Arabs (0.67%), but the disparity diminished when socioeconomic status was considered. CONCLUSIONS: This unprecedented Population study establishes a definitive rate of true pubertal gynecomastia, revealing a lower yearly incidence as compared to previous reports. The higher observed prevalence among Jewish adolescents, may be caused due to complex interactions between different influencing factors. Understanding these dynamics can aid in formulating more targeted interventions and policy strategies to address gynecomastia's affect on adolescent well-being.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10953823/

Record 413

pmcid: PMC10990311

pmid: 38576976

doi: 10.1097/MS9.0000000000001836

title: Recurrent fibroepithelial vaginal polyp in a 2-year-old girl: a case report and review of the literature.

authors

- Shehadeh MH
- Abualrub AM
- Malhes WM
- Msarweh A
- Amro W

journal: Annals of medicine and surgery (2012)

journalAbbr: Ann Med Surg (Lond)

year: 2024

pubdate: 2024 Apr

volume: 86

issue: 4

pages: 2335-2338

abstract: INTRODUCTION AND IMPORTANCE: Fibroepithelial vaginal polyps (FEPV) are rare mucosal polypoid lesions primarily found in adult women. However, FEPV in paediatric patients, especially beyond the neonatal period, is exceedingly uncommon. Documenting cases improves diagnosis and management. FEPV can mimic malignancy, posing challenges for interpretation. Early detection, treatment, and follow-up are crucial for optimal outcomes. CASE PRESENTATION: The authors present the case of a 2-year-old girl with a recurrent FEPV. The patient had a painless, reddish, smooth, soft, rubbery polypoid vaginal mass measuring 2.3 × 1.5 cm. Physical examination revealed no concurrent systemic issues. Surgical excision of the polyps confirmed them as fibroepithelial polyps. Despite previous excisions, the polyps recurred within a month. The patient's postoperative recovery was uneventful, and subsequent follow-up showed no recurrence. CLINICAL DISCUSSION: FEPV presents as mucosal polypoid lesions with a connective tissue core covered by benign squamous epithelium. It is rare before menarche and after menopause. Although the pathophysiology remains unclear, hormonal factors and local injuries may contribute. FEPV is usually asymptomatic but may cause pressure, obstruction, bleeding, or discharge. Differential diagnosis includes vaginal connective tissue malignancies. The diagnosis was confirmed by surgical excision and histopathology. Complete excision is crucial for preventing recurrence. CONCLUSION: This case report highlights recurrent FEPV in a 2-year-old girl. Despite previous excisions, polyps recurred, emphasizing the need for complete excision. Documenting cases will enhance our understanding. Further research is needed to elucidate the pathogenesis of paediatric FEPV. Early detection, treatment, and follow-up are essential for optimal management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10990311/

Record 414

pmcid: PMC11162529

pmid: 38855402

doi: 10.51866/lte.46lr

title: Reply letter to: If SARS-CoV-2 vaccination is blamed for Parsonage-Turner syndrome, neurosurgical neurolysis is not indicated.

authors

- Yeoh ZY
- Ramdzan SN

journal: Malaysian family physician : the official journal of the Academy of Family Physicians of Malaysia

journalAbbr: Malays Fam Physician

year: 2024

pubdate: 2024

volume: 19

issue:

pages: 34

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11162529/

Record 415

pmcid: PMC10936915

pmid: 38224575

doi: 10.5935/1518-0557.20230061

title: Restructuring healthcare services, routines and procedures on reproductive medicine based on respect for differences.

authors

- Antunes Junior N
- Galuppo AG
- Soares JB
- Glina S

journal: JBRA assisted reproduction

journalAbbr: JBRA Assist Reprod

year: 2024

pubdate: 2024 Feb 26

volume: 28

issue: 1

pages: 96-102

abstract: Although the term homosexuality was removed from the International Classification of Diseases and trans identities from mental disorders, these classifications promote the pathologizing of homosexuality. The direct consequence is discrimination, which adds to the difficulty in carrying out accurate information related to the LGBT population and makes it very difficult to organize public policies suited to their needs. An important issue is related to the limited access of that population to assisted reproduction techniques, when compared to traditional families. The desire for same sex couples and transgender persons to have biological children is reportedly the same as for cisgender persons, but parenthood can be a much greater endeavor both medically and psychologically for them. The right to health includes freedom to control one's health and body, including sexual and reproductive issues. Despite these difficulties, we are living in a period of great social progress that increases access to assisted reproduction among novel patient populations. With legalization of gay marriage, individuals and couples who identify as lesbian, gay, bisexual and transgender, may seek to begin or expand their families with assisted reproduction technologies. Therefore, the aim of this review was to assist in the restructuring of healthcare services, routines and procedures, mainly related to reproductive medicine, in order to promote changes in values based on respect for differences. In conclusion, the healthcare personnel of fertility centers should undergo specific training and preparations to meet the specific demands of the LGBT patient population and to overcome communication barriers.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10936915/

Record 416

pmcid: PMC11541300

pmid: 38459382

doi: 10.1007/s00266-024-03922-5

title: Retrospective Study on Optimizing Breast Augmentation Outcomes in Transgender Patients: A Comprehensive Analysis of Tumescent Local Anesthesia Technique.

authors

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- Arrica G
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journal: Aesthetic plastic surgery

journalAbbr: Aesthetic Plast Surg

year: 2024

pubdate: 2024 Oct

volume: 48

issue: 19

pages: 3817-3824

abstract: BACKGROUND: Tumescent local anesthesia (TLA) involves the infusion of a saline solution containing lidocaine and epinephrine into tissues to achieve localized anesthesia and vasoconstriction. While the use of TLA in sub-glandular augmentation mammoplasty has been documented, we present a modified TLA approach for primary sub-muscular breast augmentation in transgender patients based on our experience over the past years. METHODS: Between the years 2014 and 2021, we performed primary sub-muscular breast augmentation on 20 transgender patients under TLA and conscious sedation. The tumescent solution consisted of 25 mL of 2% lidocaine, 8 mEq of sodium bicarbonate, and 1 mL of epinephrine (1 mg/1 mL) in 1000 mL of 0.9% saline solution. Initially, the solution was infiltrated between the pectoral fascia and the mammary gland, and subsequently, during the surgery, under the pectoralis major muscle. RESULTS: The average volume of tumescent solution infiltrated during TLA was 740 mL per breast. There were no reports of adrenaline or lidocaine toxicity, and no cases required a conversion to general anesthesia. Patients experienced no pain or discomfort during the preoperative infiltration or surgical procedure. Reoperations due to short-term complications never occurred. We observed a major complication rate of 5%, represented by 1 hematoma. Long-term complications comprised one case of implant dislocation and one occurrence of dystrophic scar formation. No cases of capsular contracture needing reoperation, asymmetry, and implant rupture occurred. In total, one individual (5%) requested larger implants. Follow-up time ranged from 30 days to 1 years. CONCLUSIONS: Overall, augmentation mammaplasty is a valuable choice for transgender women aiming to enhance their feminine characteristics and alleviate gender dysphoria. It is imperative for patients to conduct thorough research, grasp the potential pros and cons, and consult experienced healthcare professionals in transgender care. Additionally, tumescent local anesthesia (TLA) has proven to be a safe and efficient method for sub-muscular breast augmentation, providing effective pain control with minimal postoperative complications, resulting in high patient satisfaction. LEVEL OF EVIDENCE IV: This journal requires that authors assign a level of evidence to each article. For a full description of these Evidence-Based Medicine ratings, please refer to the Table of Contents or the online Instructions to Authors www.springer.com/00266 .

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11541300/

Record 417

pmcid: PMC11189279

pmid: 38911112

doi: 10.4103/ijem.ijem_146_23

title: Role of Inhibin B, AMH, GnRHa Test and HCG Stimulation Test to Distinguish Isolated Hypogonadotropic Hypogonadism (IHH) from Constitutional Delay in Growth and Puberty (CDGP).

authors

- Sahoo BK
- Kumar PR

- Pattanaik SR
- Dash DK
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journal: Indian journal of endocrinology and metabolism

journalAbbr: Indian J Endocrinol Metab

year: 2024

pubdate: 2024 Mar-Apr

volume: 28

issue: 2

pages: 153-159

abstract: INTRODUCTION: This study aimed to distinguish isolated hypogonadotropic hypogonadism (IHH) from constitutional delay in growth and puberty (CDGP) by various hormonal tests in both sexes. METHODS: Boys with testicular volume (TV) <4 ml (14-18 years) and girls with breast B 1 stage (13-18 years) were enrolled in this study. A detailed history, clinical examination and hormonal analysis including basal luteinising hormone (LH), follicle-stimulating hormone (FSH), inhibin B, anti-Mullerian hormone (AMH), testosterone (boys), oestradiol (girls), triptorelin stimulation test and 3-day human chorionic gonadotropin (HCG) stimulation test (boys) were performed. All patients were followed for 1.5 years or till 18 years of age. Receiver operating characteristic (ROC) curve analysis was performed to determine the optimal cut-offs with sensitivity, specificity, positive predictive value (PPV) and negative predictive value (NPV) for various hormones to distinguish IHH from CDGP. RESULTS: Of 34 children (male: 22 and female: 12), CDGP and IHH were diagnosed in 21 and 13 children, respectively. 4 hours post-triptorelin LH had the highest sensitivity (100%) and specificity (100%) for identifying IHH in both sexes. Basal inhibin B had good sensitivity (male: 85.7% and female: 83.8%) and specificity (male: 93.3% and female: 100%) for diagnosing IHH. 24 hours post-triptorelin testosterone (<34.5 ng/dl), day 4 post-HCG testosterone (<99.7 ng/dl) and 24 hours post-triptorelin oestradiol (<31.63 pg/ml) had reasonable sensitivity and specificity for identifying IHH. Basal LH, FSH and AMH were poor discriminators for IHH in both sexes. CONCLUSION: The best indicator was post-triptorelin 4-hour LH followed by inhibin B, which had a reasonable diagnostic utility to distinguish IHH from CDGP in both boys and girls.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11189279/

Record 418

pmcid: PMC11203090

pmid: 38921261

doi: 10.3390/clinpract14030083

title: Safety of Progestogen Hormonal Contraceptive Methods during Lactation: An Overview.

authors

- Chmaj-Wierzchowska K
- Wszołek K
- Tomczyk K
- Wilczak M

journal: Clinics and practice

journalAbbr: Clin Pract

year: 2024

pubdate: 2024 Jun 4

volume: 14

issue: 3

pages: 1054-1064

abstract: BACKGROUND: Breastfeeding is a process for not only nourishing infants but also for building a unique emotional bond between mother and child. Therefore, the ideal contraception during lactation should not affect lactation (milk composition, milk volume) and offspring development. OBJECTIVES: This study aims to analyze the literature on the safety of progestogen hormonal contraceptive methods during lactation. METHODS: We conducted a thorough search across various databases, including the National Library of Medicine (PubMed), and the Cochrane Database, Drugs and Lactation Database (LactMed). Our search utilized specific phrases such as: "lactation" and "breastfeeding" and "oral contraception" with "drospirenone" or "desogestrel", with "subcutaneous etonogestrel implant" or "etonogestrel implant", with "levonorgestrel-releasing intrauterine system", and "emergency contraception", with "levonorgestrel" or "ulipristal acetate". CONCLUSIONS: Based on published scientific reports, progestogen hormonal contraceptives can be considered a relatively safe solution for women desiring to continue feeding their infant with their milk while using hormonal contraception. It is important to seek guidance on selecting the best contraception method based on the latest medical knowledge, tailored to the individual needs and clinical circumstances of each woman and place of residence. A woman should always be informed of the potential risks of such a treatment and then allowed to make her own decision based on the knowledge received from a specialist.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11203090/

Record 419

pmcid: PMC11557297

pmid: 39544189

doi: 10.14440/jbm.2024.0019

title: Side effects of prostate cancer therapies and potential management.

authors

- Xiao J
- Zhang M
- Wu D

journal: Journal of biological methods

journalAbbr: J Biol Methods

year: 2024

pubdate: 2024

volume: 11

issue: 3

pages: e99010018

abstract: Prostate cancer (PCa) remains a significant health challenge, necessitating diverse therapeutic interventions to manage the disease effectively. While these treatments offer promising outcomes, they are often accompanied by a range of side effects that can impact patient quality of life and treatment compliance. This review provides an overview of the common side effects associated with various PCa therapies, including prostatectomy, radiation therapy, thermal therapy, hormone therapy, chemotherapy, and targeted drug therapy, among others. We summarized and discussed the reported side effects encompassing ureteral problems, sexual issues, gastrointestinal symptoms, fatigue, anemia, thrombocytopenia, hematologic abnormalities, nausea, vomiting, and liver enzyme elevation. Specific managements, such as personalized treatment plans, proactive symptom monitoring, supportive care interventions, and hematological assessments, are crucial in mitigating these side effects and optimizing treatment outcomes. By prioritizing patient-centered care and tailored interventions, health-care providers can enhance treatment efficacy and improve the overall well-being of individuals undergoing PCa therapies.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11557297/

Record 420

pmcid: PMC10918796

pmid: 38463254

doi: 10.1021/acsomega.3c09648

title: Study on the Efficacy and Potential Mechanism of Topical Shen Bai Hair Growing Decoction against Androgenetic Alopecia Based on Ultrahigh Performance Liquid Chromatography Quadrupole Time-of-Flight Mass Spectrometry and RNA-seq.

authors

- Li M
- Zhang X
- Wang Y
- Xiang B
- Liu Z
- Zhang W
- Liu X
- Guo R

journal: ACS omega

journalAbbr: ACS Omega

year: 2024

pubdate: 2024 Mar 5

volume: 9

issue: 9

pages: 10834-10851

abstract: Androgenetic alopecia (AGA) is a major problem that can happen to people of all ages, leading to psychological problems, such as anxiety and depression. Topical Shen Bai hair growing decoction (TSBHGD) is based on the pathogenesis of AGA, combined with Traditional Chinese Medicine theory, improved by the Tianjin Academy of Traditional Chinese Medicine Affiliated Hospital according to its clinical treatment experience. This study was designed to demonstrate the therapeutic efficacy of TSBHGD against AGA, analyze the chemical components of TSBHGD as well as the skin-retained and blood-retained components in mice after topical administration of TSBHGD, and clarify the mechanism of its therapeutic efficacy. It was demonstrated that TSBHGD could suppress TNF-α and IL-6 levels and improve pathological phenomena such as hair loss, reduced follicle density, and dermal thickness caused by testosterone solution. Totally 35 components were identified in TSBHGD extracts, 12 skin-retained components were identified in drug-containing skin, and 7 blood-retained components were identified in drug-containing plasma, according to ultrahigh performance liquid chromatography quadrupole time-of-flight mass spectrometry. Transcriptomic sequencing revealed that some of the genes in AGA mice had altered expression patterns, which could be reversed by TSBHGD. Through network pharmacology analysis, it was found that TSBHGD mainly regulated eight signaling pathways, among which the apoptosis signaling pathway ranked first with a significance of 0.00149. Finally, both Bcl-2 and Caspase family proteins in the apoptosis signaling pathway were examined by Western blot. It was confirmed that TSBHGD could inhibit the apoptosis level in AGA mice's skin tissue to exert an anti-AGA effect. This will facilitate the development of new-generation herbal compound formulas with precise efficacy and provide novel ideas for AGA therapy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10918796/

Record 421

pmcid: PMC11106200

pmid: 38228983

doi: 10.1007/s10508-023-02787-0

title: Subjective Experiences of Pregnancy, Delivery, and Nursing in Transgender Men and Non-Binary Individuals: A Qualitative Analysis of Gender and Mental Health Concerns.

authors

- Falck FAOK
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journal: Archives of sexual behavior

journalAbbr: Arch Sex Behav

year: 2024

pubdate: 2024 May

volume: 53

issue: 5

pages: 1981-2002

abstract: Studies of how gender-diverse individuals experience pregnancy, childbirth, and nursing remain few, mainly focus on the US and contain scarce information about mental health concerns peri-partum. This hinders informed reproductive health decisions and counseling. We used in-depth interviews to examine how gestational gender-diverse individuals in Sweden experience the process of planning and undergoing pregnancy, delivery, and nursing. In total, 12 participants, identifying on the masculine side of the gender spectrum or as non-binary, who had attended Swedish antenatal care and delivered a live birth, were included in the study. Data were analyzed using qualitative thematic content analysis. The analysis resulted in one overarching theme: sustaining gender congruence during pregnancy and three main categories: (1) considering pregnancy; (2) undergoing pregnancy and childbirth; and (3) postnatal reflections. The association between childbearing and being regarded as female permeated narratives. Participants renegotiated the feminine connotations of pregnancy, accessed gender-affirming treatment, and concealed their pregnancy to safeguard their gender congruence. Mis-gendering and breast enlargement triggered gender dysphoria. Social judgment, loneliness, information shortages, hormonal influence and cessation of testosterone increased gender dysphoria and strained their mental health. Depression exacerbated gender dysphoria and made it harder to claim one's gender identity. Dissociation was used to handle a feminized body, vaginal delivery, and nursing. Pregnancy was easier to envision and handle after masculinizing gender-affirming treatments. The results deepen the understanding of gender dysphoria and may be used to inform reproductive counseling and healthcare development. Research outcomes on mental health concerns provide a basis for further research.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11106200/

Record 422

pmcid: PMC11661758

pmid: 39712371

doi: 10.1097/GOX.0000000000006399

title: The "Pfizer Boob Job": A Case of Unexplained Gigantomastia.

authors

- Balogun OT
- Osei B

- Armstrong KA

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2024

pubdate: 2024 Dec

volume: 12

issue: 12

pages: e6399

abstract: True parenchymal breast changes following COVID-19 vaccination are exceedingly rare. Equally rare are incidences of pseudoangiomatous stromal hyperplasia (PASH)-associated gigantomastia. The patient in this case report presented with both occurrences and experienced massive breast enlargement 1 week postadministration of the Pfizer/BioNTech COVID-19 vaccine, which worsened after her second dose. Definitive treatment was achieved through reduction mammoplasty, with the final pathology report confirming uniformly dense terminal duct lobular units with interspersed PASH units. The association between the COVID-19 vaccine, PASH, and breast hypertrophy warrants further investigation to comprehend the spectrum of reactions to the vaccine.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11661758/

Record 423

pmcid: PMC11136523

pmid: 38813345

doi: 10.7759/cureus.59306

title: The Case of the Positive Pregnancy Pretender: A Potential Cause of Erroneous Test Results in Active-Duty Personnel.

authors

- Corless JM
- Glendening JM
- Burtson K

journal: Cureus

journalAbbr: Cureus

year: 2024

pubdate: 2024 Apr

volume: 16

issue: 4

pages: e59306

abstract: The detection of pregnancy is common among those who participate in the care of reproductive-age females. This is especially true in the medical care of active-duty personnel in the armed forces. Considering the impact of a positive urine pregnancy test in this population, it is important to recognize the possibility of false-positive results and their causes. In this case, we explore a false-positive urine pregnancy test due to injectable positive beta-human chorionic gonadotropin (beta-hCG) supplementation used for weight loss. This report concludes that the use of exogenous beta-hCG by physicians and other clinicians should be avoided. Additionally, clinicians should be aware of its use in the community and its possible effect on laboratory testing used to evaluate for pregnancy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11136523/

Record 424

pmcid: PMC10862426

pmid: 38356806

doi: 10.12669/pjms.40.3.7447

title: The clinical value of serum-related indexes in differentiating simple premature thelarche from idiopathic central precocious puberty.

authors

- Cui HJ
- Liu P
- Liu XG
- Du WZ
- Wang X

journal: Pakistan journal of medical sciences

journalAbbr: Pak J Med Sci

year: 2024

pubdate: 2024 Jan-Feb

volume: 40

issue: 3Part-II

pages: 467-472

abstract: OBJECTIVE: To explore the changes of serum-related indexes at different time points, so as to identify the critical time of converting from simple premature thelarche (PT) to idiopathic central precocious puberty (ICPP). METHODS: This is a retrospective study. The subjects of the study were 50 girls with PT who were admitted to the Children's Hospital of Hebei Province from January 2019 to September 2020. The enrolled 50 children were divided into the conversion group(n=12) and the non-conversion group(n=38) according to whether PT was converted into ICPP during follow-up. Furthermore, the levels of serum-related indexes and uterine and ovarian volumes were compared after the diagnosis of PT. RESULTS: The IGF-1 and IGFBP-3 levels of children in the conversion group began to change significantly from six months after the diagnosis, with statistically significant differences when compared with the levels of children at the initial diagnosis, three months and those of the non-conversion group at the same time points (p< 0.05). The levels of vitamin-D, DHEA and leptin began to change significantly at nine months after the diagnosis (p< 0.05). Besides, uterine and ovarian volumes in the conversion group began to increase significantly six months after the diagnosis, with statistically significant differences when compared with those in the non-conversion group (p< 0.05). CONCLUSION: Findings in our study suggest that regular monitoring of vitamin-D, IGF-1, IGFBP-3, DHEA and leptin levels, and uterine and ovarian volumes can predict the conversion from PT to ICPP at an early stage.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10862426/

Record 425

pmcid: PMC10847246

pmid: 38327564

doi: 10.3389/fendo.2024.1314752

title: The effect of gonadoliberin analog treatment in precocious puberty on polycystic ovarian syndrome prevalence in adulthood.

authors

- Orszulak D
- Niziński K

- Bil A
- Gawlik A
- Ziora K
- Drosdzol-Cop A

journal: Frontiers in endocrinology

journalAbbr: Front Endocrinol (Lausanne)

year: 2024

pubdate: 2024

volume: 15

issue:

pages: 1314752

abstract: UNLABELLED: Precocious puberty is diagnosed when pubertal characteristics appear before the age of 8 years in females. The most common form is gonadotropin-dependent, called axial. The primary method of treatment is administration of gonadotrophin-releasing hormone analogues (GnRHa). The aim of the study was to verify hypothesis that GnRHa therapy in the childhood may be of additive risk factor for polycystic ovary syndrome (PCOS) in adulthood. MATERIAL AND METHODS: The study group consists of 24 women (median age 22 88 years, median BMI 23.5) treated with GnRHa for central precocious puberty in childhood. The control group includes 40 women (median age 23 years, median BMI 25.6) diagnosed with isolated premature thelarche and not using GnRHa in the childhood. Anthropometric measurements, ultrasound examination of minor pelvis and hormonal profile were performed. PCOS diagnosis was based on Rotterdam criteria. RESULTS: The study confirmed a higher prevalence of PCOS in the study group (50%) than in the control group (10%); p=0.0006. Significant, linear correlation between free testosterone levels and ovarian size was found in the study group (R=0.45 p= 0.03). CONCLUSIONS: GnRHa therapy during childhood may have a potential influence on incidence of PCOS in the adulthood. Therefore, in this group of patients long-term follow-up focused on screening for PCOS would seem beneficial.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10847246/

Record 426

pmcid: PMC11365678

pmid: 39220751

doi: 10.1097/GOX.0000000000006099

title: The Efficacy and Safety Assessment of Sequential Liposuction Modified Double Ring Areola Incision in Moderate and Severe Breast Ptosis Correction.

authors

- Tan Y
- Yao C
- Chen B
- Yuan X

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2024

pubdate: 2024 Aug

volume: 12

issue: 8

pages: e6099

abstract: BACKGROUND: A total of 42 cases of moderate and severe breast ptosis were treated with sequential liposuction modified double ring areola incision surgery (experimental group), and 45 cases were treated with traditional double ring areola incision alone surgery (control group). METHODS: They were followed up for 6 months after the operation. The scar formation and breast ptosis repair were observed, and postoperative complications and patient satisfaction were monitored. RESULTS: Better results such as improved postoperative breast appearance were achieved in the sequential liposuction modified double ring areola incision group compared with the double ring areola incision alone group. After treatment, the distance between the midpoint of the inframammary fold and the nipple, the distance between the suprasternal fossa, and the nipple of the two groups were lower than those before treatment. The distance between the midpoint of the inframammary fold and the nipple showed no difference between the two experimental groups. The distance between the suprasternal fossa and the nipple of the experimental group were lower compared with the control group. Lower complication rates and higher satisfaction rates were observed in patients treated with sequential liposuction modified double ring areola incision. The patients in the experimental group were more satisfied with breast symmetry, breast shape, and breast size than those in the control group. CONCLUSIONS: The treatment with sequential liposuction modified double ring areola incision was effective and safe compared with double ring areola incision alone. It could improve the breast shape of patients, improve the satisfaction of patients, and reduce the occurrence of complications.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11365678/

Record 427

pmcid: PMC10845625

pmid: 38317236

doi: 10.1186/s13010-024-00151-1

title: The ethical foundations of patient-centered care in aesthetic medicine.

authors

- da Prato EB
- Cartier H
- Margara A
- Molina B
- Tateo A
- Grimolizzi F
- Spagnolo AG

journal: Philosophy, ethics, and humanities in medicine : PEHM

journalAbbr: Philos Ethics Humanit Med

year: 2024

pubdate: 2024 Feb 5

volume: 19

issue: 1

pages: 1

abstract: This article addresses some critical aspects of the relationship between aesthetic medicine (AM) and ethics and proposes a possible deontological ethical line to pursue based on current practices. The role of AM has always been controversial and suffers from unclear practical and moral boundaries, even within academic settings, since it aims to improve the appearance of individuals, not to cure a disease. Today, it is essential and pertinent to discuss these issues, as AM specialists are dealing with a growing and increasingly demanding patient population that has undergone profound evolution in recent years. Current challenges within the field of AM include a lack of global uniformity concerning the education of AM specialists, an increasing number of

physicians practicing AM with diverse training backgrounds, the spread of AM being practiced outside of medical practice or hospital settings, and the influence of social media where the success is modelled and dictated by the identification of a youthful appearance). By the field of action enriched by technologies that aim not only at enhancement per se but also at the preservation and regeneration of tissues, it is necessary to establish an active multidisciplinary discussion on the definition of shared ethical limits. This discussion would allow AM to fully reclaim its identity as a specialty that aims to improve patient well-being whilst maintaining respect for patient aesthetic harmony, the expertise of specialists who practice AM, the essential role of safety, and awareness of the importance of a confidential doctor-patient relationship.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10845625/

Record 428

pmcid: PMC11392988

pmid: 39080057

doi: 10.1007/s00404-024-07648-2

title: The impact of pregnancy on sexual functioning in Polish women.

authors

- Szymańska E
- Kisielewski R
- Kisielewska L
- Tomaszewski J

journal: Archives of gynecology and obstetrics

journalAbbr: Arch Gynecol Obstet

year: 2024

pubdate: 2024 Oct

volume: 310

issue: 4

pages: 2133-2140

abstract: INTRODUCTION: Sexual life of pregnant women alters during pregnancy due to the physiologic,' anatomic and hormonal changes in her body. Therefore, the aim of this study was to evaluate female sexual functioning after becoming pregnant. PATIENTS AND METHODS: A prospective survey study including 148 pregnant women. An anonymous questionnaire including 60 inquiries concerning intimate relationship before and during pregnancy was performed. The following statistical test were used: Chi-square test of independence, Spearman's rank correlation coefficient and Wilcoxon test. The significance level of $p = 0.05$ was assumed. RESULTS: Most of the respondents were between 31 and 40 years old (55%). Majority of them were married (86%). During pregnancy, slightly more than half of women had a moderate need for sexual intercourse (51%), a large percentage of them had a low need (32%), a high need for intercourse was declared by 17% of women. The correlation analysis showed a statistically significant relationship between women's education and the need for sex before pregnancy ($p = 0.049$). Respondents with higher degrees of education more often felt the need for intercourse before pregnancy. No correlation was found between education and the need for intercourse after pregnancy ($p = 0.107$). After becoming pregnant, 51% of women had less need for intercourse, 7% more, and 42% the same as before pregnancy, and these differences were statistically significant ($p < 0.001$). Also, a decreased satisfaction with sexual intercourse was reported during pregnancy ($p < 0.001$). After getting pregnant, the average number of intercourses decreased in majority (71%) of respondents, and differences in the number of intercourses before and during pregnancy were statistically significant ($p < 0.001$). CONCLUSION: Pregnancy has significant impact on woman's sexuality. After becoming pregnant majority of women declare less need for sexual intercourses, decreased number of intercourses with less satisfaction.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11392988/

Record 429

pmcid: PMC11491692

pmid: 39439736

doi: 10.18502/jfrh.v18i3.16659

title: The Most Common Side Effects, Concerns and Discontinuation Patterns of Oral Contraceptive Pills Among Polish Women: A Cross-Sectional Study.

authors

- Karpowicz N
- Mączka K
- Skoczylas A
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- Kunicki M
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journal: Journal of family & reproductive health

journalAbbr: J Family Reprod Health

year: 2024

pubdate: 2024 Sep

volume: 18

issue: 3

pages: 170-179

abstract: OBJECTIVE: Over the decade, variety and effectiveness of contraception methods have greatly improved, resulting in increased popularity of oral contraceptive pills (OCPs). However, hormonal contraception carries the risk of multiple side effects. The lack of sufficient knowledge often leads to the development of health concerns, which can affect patient's adherence. The aim of our research was to describe a side effect profile of OCPs and assess the most frequent concerns and discontinuation reasons in Polish women. The survey included questions regarding OCPs utilization patterns, side effects, health concerns and attitudes of the responders. MATERIALS AND METHODS: This cross-sectional study was based on a survey, which was distributed online and open from 29 April to 15 May 2022. The survey included questions regarding OCPs utilization patterns, side effects, health concerns and attitudes of the responders. RESULTS: Out of 1699 respondents, the current OCPs intake was reported by two thirds of women and 22% had a history of using them in the past. Seventy-nine percent of all OCPs users experienced adverse effects while 9% reported having concerns about safety and potential adverse effects. Decreased libido and weight gain were the most significant reasons for ceasing hormonal contraception. Moreover, the most common concerns and the most unfavorable side effects varied in different age groups. The occurrence of anxiety depended on age and education. CONCLUSION: Healthcare professionals prescribing OCPs should provide their patients with comprehensive counseling. Understanding and addressing concerns of young women can improve their compliance and reduce the number of unintended pregnancies.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11491692/

Record 430

pmcid: PMC11372348

pmid: 39233778

doi: 10.3310/nihropenres.13652.2

title: The UK Breast Cancer in Pregnancy (UKBCiP) Study. Incidence, diagnosis, management and short-term

outcomes of breast cancer first diagnosed during pregnancy in the United Kingdom: A population-based descriptive study.

authors

- Hardy C
- Brand A
- Jones J
- Knight M
- Banfield P

journal: NIHR open research

journalAbbr: NIHR Open Res

year: 2024

pubdate: 2024

volume: 4

issue:

pages: 40

abstract: BACKGROUND: The incidence of breast cancer first arising during pregnancy has been estimated in several countries to be 2.4-7.8/100,000 births, but has not been established in the United Kingdom (UK). We aimed to estimate the incidence of breast cancer diagnosed during pregnancy in the UK and to describe its management and short-term outcomes for mothers and babies. METHODS: This population-based descriptive study used the UK Obstetric Surveillance System (UKOSS). Cases were prospectively identified through monthly UKOSS mailings to all UK consultant-led maternity units. All cases of breast cancer diagnosed first during pregnancy, between October 1, 2015, and September 30, 2017, were eligible, with 84 confirmed cases analysed. Women with breast cancer diagnosed before pregnancy or with a recurrence were excluded. The primary outcomes were the incidence of breast cancer first diagnosed during pregnancy, maternal mortality, severe maternal morbidity, perinatal mortality, and severe neonatal morbidity. RESULTS: The incidence was 5.4/100,000 maternities (95% CI 4.37, 6.70). Nine women (11%) had undergone in vitro fertilisation (IVF), compared with a contemporaneously estimated 2.6% IVF pregnancies in the UK. During pregnancy, 30 women (36%) underwent surgery and 37 (44%) received chemotherapy. Three women had major maternal morbidity during pregnancy. Two women died and two perinatal deaths occurred. CONCLUSIONS: The incidence of breast cancer arising in pregnancy in the UK is similar to that reported elsewhere. The higher proportion of IVF pregnancies among affected women needs further investigation, as it may not be entirely explained by relatively advanced maternal age. With caveats, management followed that outside pregnancy, but there was considerable variation in practice. Although short-term outcomes were generally good for mothers and babies, a larger prospective study is required. Iatrogenic pre-term delivery and its associated risks to the infant can often be avoided; treatment was administered during pregnancy without evidence of harms to the infant.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11372348/

Record 431

pmcid: PMC11419685

pmid: 39319967

doi: 10.1590/0034-7167-2023-0346

title: Transgender people's knowledge about the adverse effects of cross-hormonization: challenges for nursing.

authors

- Ahmad AF
- Lemos A
- Ribeiro CR

- Araujo LM
- Corrêa VAF
- Janini JP
- Mello R
- Dantas BRSS

journal: Revista brasileira de enfermagem

journalAbbr: Rev Bras Enferm

year: 2024

pubdate: 2024

volume: 77

issue: 4

pages: e20230346

abstract: OBJECTIVES: to identify trans women's and men's knowledge about the adverse effects of cross-hormonization and understand the repercussions of hormonization practices on trans women's and men's health. METHODS: exploratory, descriptive, qualitative research, developed with 41 participants, from July 2019 to February 2020, in a trans health outpatient clinic. Thematic-categorical content analysis was used. RESULTS: from the analysis, the categories emerged: Knowledge about the adverse effects of cross-hormonization; and Cross-hormonization practices and their meaning. FINAL CONSIDERATIONS: nursing practices, based on the identification of knowledge about adverse effects and the understanding of cross-hormonization practices in trans women's and men's health, can result in more inclusive care.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11419685/

Record 432

pmcid: PMC11538920

pmid: 39451189

doi: 10.5114/pedm.2024.142585

title: Transient clitoromegaly in an extremely preterm twin infant with popliteal pterygium.

authors

- Sabolić LG
- Kovačević A
- Trtanj LA
- Morić BV
- Tumbri J

journal: Pediatric endocrinology, diabetes, and metabolism

journalAbbr: Pediatr Endocrinol Diabetes Metab

year: 2024

pubdate: 2024

volume: 30

issue: 3

pages: 163-167

abstract: Clitoromegaly can be congenital or acquired, and it is usually associated with exposure to androgen excess. Pathophysiological mechanisms responsible for transient clitoromegaly in premature female infants have not been fully elucidated. Herein, we present the case of an extremely premature female twin infant, with

an extensive web of skin on the back of the left leg, hypoplastic left labia majora, and normal clitoris appearance at birth. At the age of 48 days, clitoral enlargement was observed. Significantly elevated levels of gonadotropins, testosterone, and dehydroepiandrosterone sulphate (DHEAS) were recorded. 17-hydroxyprogesterone (17OHP) was unremarkable, and anti-Müllerian hormone (AMH) was low, in accordance with normal female karyotype. Ovaries were not visualised ultrasonographically. During the following weeks, gradual normalisation of gonadotropin, testosterone, and DHEAS levels was accompanied by regression of clitoromegaly. As described in this case, transient clitoral enlargement may appear in extremely premature female infants due to transitory elevated androgens of ovarian and adrenal origin.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11538920/

Record 433

pmcid: PMC11246854

pmid: 39011455

doi: 10.3389/fmed.2024.1364089

title: Unilateral severe gynecomastia in a 14 year-old adolescent with neurofibromatosis type 1 undergoing endoscopic mastectomy: a case report.

authors

- Shang F
- Xi B
- Qiu D
- Chen X
- Wang Y
- He M
- Liu B
- Zhao Z

journal: Frontiers in medicine

journalAbbr: Front Med (Lausanne)

year: 2024

pubdate: 2024

volume: 11

issue:

pages: 1364089

abstract: Gynecomastia can be caused by neurofibromas but has rarely been reported. The present case report describes the clinical appearance, diagnosis, and therapy of a rare combination of a 14 year-old adolescent male unilateral severe gynecomastia with NF-1 neurofibromatosis. In this particular case, we successfully performed minimally invasive surgery using endoscopic mastectomy, which not only resulted in a satisfactory appearance but also confirmed the presence of neurofibroma type 1 by detecting typical immunohistochemical indicators associated with the disease. Additionally, we analyzed the gene responsible for the disease, c.1431del: p. F477Lfs*21, based on the patient's family history.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11246854/

Record 434

pmcid: PMC11570433

pmid: 39559668

doi: 10.7759/cureus.71809

title: Unilateral Virginal Breast Hypertrophy: A Report of a Rare Condition With an Unknown Etiology.

authors

- Kirdat Patil PP
- Yadav P
- Rangankar V
- C Buch A

journal: Cureus

journalAbbr: Cureus

year: 2024

pubdate: 2024 Oct

volume: 16

issue: 10

pages: e71809

abstract: Unilateral virginal hypertrophy of the breast is a rare condition with an unknown etiology, though it is thought to result from hypersensitivity of the end organs to normal levels of gonadal hormones. We present the case of a 17-year-old girl who initially showed gradual growth after thelarche; however, in the last three to four months, there was a rapid increase in size, leading to psychological and physical problems that also impacted her social life. A clinical examination revealed that her left breast was markedly enlarged compared to the right, and she exhibited a drooping posture. Radiological investigations and a true-cut biopsy were performed to detect any underlying pathology, which revealed normocellular stroma consistent with virginal breast hypertrophy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11570433/

Record 435

pmcid: PMC11331716

pmid: 39161564

doi: 10.1016/j.radcr.2024.06.072

title: Unusual radiologic imaging in juvenile granulosa cell tumor with precocious puberty: A unilocular cyst.

authors

- Afriliani N
- Wulanhandarini T

journal: Radiology case reports

journalAbbr: Radiol Case Rep

year: 2024

pubdate: 2024 Oct

volume: 19

issue: 10

pages: 4283-4288

abstract: Juvenile Granulosa Cell Tumor (JGCT) represents 5% of all granulosa cell cancers. Precocious puberty is a frequent feature of this tumor. A 2-year and 2-month-old girl was referred with a diagnosis of suspected ovarian cancer, dysfunctional uterine bleeding, and precocious puberty. Radiologic examination revealed the following: Abdominal ultrasonography showed a solitary anechoic cystic lesion in the pelvic cavity. MRI confirmed the existence of solid components on its walls. JGCT was then confirmed using

immunohistochemistry (IHC) markers. JGCT, along with adult granulosa cell tumors (AGCT) are subgroups of granulosa cell tumors (GCTs), which are part of pure sex cord tumors. The 2 forms share imaging findings due to their comparable gross appearance. GCTs require diagnostic imaging tests to distinguish them from other ovarian tumors. Two ultrasound patterns can be identified GCTs, and MRI showed that GCTs are more heterogeneous than other sex-cord stromal tumors (OSCs). In our case, the imaging characteristics for juvenile granulosa cell tumors were nonspecific and these tumors cannot be reliably distinguished from other ovarian neoplasms based on imaging alone. Although GCTs have imaging characteristics that can help to distinguish them from other tumors, confirmation by histopathology and IHC is still mandatory, especially in cases with nonspecific radiological features.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11331716/

Record 436

pmcid: PMC10966188

pmid: 38513413

doi: 10.1016/j.ijscr.2024.109546

title: Vaginal bleeding imitated rape in a 6-year old girl, a case report about granulosa cell tumor as a reason of peripheral precocious puberty.

authors

- Amirkashani D
- Nasiri SJ
- Dadakhani S
- Mortazavi N
- Khoshkbarforoushan M

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2024

pubdate: 2024 Apr

volume: 117

issue:

pages: 109546

abstract: INTRODUCTION: Although female victims of sexual child abuse present with symptoms such as local pain and vaginal bleeding, however, before any definitive diagnosis a comprehensive physical examination along with a detailed history related to vaginal bleeding should be taken from the patient. Undoubtedly, we must not forget that only one of the causes of vaginal bleeding is rape. Therefore, before making a final diagnosis, other causes of this symptom must be carefully examined. CASE PRESENTATION: The patient was a 6-years-old female who was hospitalized for notable generalized abdominal distention, acute lower abdomen pain associated with nausea and mild fever lasting 5 days progressively worsening, thelarche and vaginal bleeding. Ultrasound examination showed that multilocular-solid masses located in right side of abdomen which led to surgery and mass excision. Histopathology diagnosis was a juvenile granulosa cell tumor of the ovary. DISCUSSION: Among the various causes of peripheral premature puberty, granulosa cell tumor (GCT) is rare but very important. Since in the two age groups - prepuberty and menopause - we don't expect to see vaginal bleeding, the occurrence of this disorder especially in association with breast enlargement in prepubertal group, need to appropriate imaging including pelvic ultrasound and bone age determination also laboratory data such as level of sex hormones and tumor markers to avoid misdiagnosis. CONCLUSION: We report the case of a granulosa cell tumor patient with vaginal bleeding that a complete history and examination provides the right path to a diagnosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10966188/

Record 437

pmcid: PMC11904172

pmid: 40092875

doi: 10.6026/9732063002001317

title: Van Wyk-Grumbach syndrome and its clinical heterogeneity: A case report.

authors

- Jain K
- Honarius SS
- Reji R
- Sangoi R
- Mutha S
- Wanjari M

journal: Bioinformation

journalAbbr: Bioinformation

year: 2024

pubdate: 2024

volume: 20

issue: 10

pages: 1317-1320

abstract: This case report presents a 6-year-old female with Van Wyk-Grumbach syndrome, highlighting its astounding clinical presentations and the challenges in its diagnosis and management. Although the disease is rare, Van Wyk-Grumbach Syndrome can have a devastating and serious impact in young boys and girls by affecting their hypothalamic-pituitary-gonadal axis. The enhancement in the medical knowledge has shown promising results in the complete treatment of the condition and superior diagnostic accuracy with suitable and necessary interventions. Treatment of Van Wyk-Grumbach Syndrome remains solely through the hormone replacement therapy of thyroid, which usually leads to complete relief of symptoms. This case highlights the unusual clinical presentation of a young female patient with Van Wyk-Grumbach Syndrome, challenging the existing diagnostic profiles and highlighting the need for personalized assessment for the patient. Since the diagnosis of this syndrome is based on the USG, MRI, X-Ray findings, enhancement in these imagining techniques will lead to increased accuracy and reduced delay in its diagnosis. This case aims to enhance the understanding of Van Wyk-Grumbach Syndrome, its medical heterogeneity and the need to advance its diagnostic and interventional prospects.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11904172/

Record 438

pmcid: PMC10730739

pmid: 38039566

doi: 10.1016/j.ijscr.2023.109097

title: Van Wyk-Grumbach syndrome: The importance of thyroid function tests in a child presenting with multicystic ovaries.

authors

- Elouali A
- Ouerradi N
- Benhaddou H

- Kamaoui I
- Rkain M
- Babakhouya A

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2024

pubdate: 2024 Jan

volume: 114

issue:

pages: 109097

abstract: INTRODUCTION: Van Wyk-Grumbach syndrome (VWGS) is a rare presentation of long-standing hypothyroidism, characterized by delayed bone age, enlarged bilateral multicystic ovaries, and isosexual precocious puberty. CASE PRESENTATION: We report here the clinical details of the case of Van Wyk Grumbach Syndrome in an 11-year-old girl who was first presented to the pediatric surgeon for hemoperitoneum. The patient underwent an emergency exploratory laparotomy, which showed enlarged ovaries with multiple cysts that were bleeding. She also had severe hypothyroidism, hyperprolactinemia, hyperestrogenism, pituitary adenoma, and delayed bone age. Significant improvement in symptoms was noted after thyroid hormone replacement therapy. IMPORTANCE AND CONCLUSION: This case highlights that it is crucial to consider thyroid assessment in a patient with multicystic ovaries to avoid misdiagnosis, unnecessary investigations for malignancy and/or surgical intervention, and possible complications.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10730739/

Record 439

pmcid: PMC11169950

pmid: 38873631

doi: 10.1016/j.eclinm.2024.102657

title: Waiting for transgender care and its effects on health and equality: a mixed-methods population study in the Netherlands.

authors

- van de Grift TC
- Martens C
- van Ginneken L
- Mullender MG

journal: EClinicalMedicine

journalAbbr: EClinicalMedicine

year: 2024

pubdate: 2024 Jul

volume: 73

issue:

pages: 102657

abstract: BACKGROUND: Access to gender-affirming medical care has life-saving effects on transgender and gender diverse (TGD) individuals. An increasing number of TGD individuals seek care which led to waiting times of years. We assessed the duration and effects of excessive waiting for gender-affirming care. METHODS: In this mixed-methods cross-sectional population study, conducted in the Netherlands between June and August

2019, all TGD individuals with experiences with receiving/applying for gender-affirming medical care could participate. Data of participants waiting for care was primarily analyzed. Demographic characteristics and requested care (multiple-choice questions) and the effects of waiting on health were surveyed (open-ended questions). Descriptive statistics were calculated per treatment, and differences in participant characteristics between those waiting for and already in care were assessed through Chi squared tests and post-hoc testing of residuals. Open text answers were thematically analyzed. FINDINGS: Of all 975 participants, 431 (44.2%) waited for gender-affirming medical care. For 10 out of 20 treatment modalities, the median waiting period was more than a year. Participants already in care were on average higher educated and less-frequently non-binary. Subjective effects of waiting were (very) negative for most treatments. Qualitative findings showed that long waiting was associated with a range of psychosocial distress, poorer health, increasing healthcare consumption, and increased inequality between TGD individuals. INTERPRETATION: Findings confirm that TGD individuals encounter long waiting times in multiple stages of their gender-affirming medical care. Waiting affects both physical and psychosocial health. Individual resilience and resources can mitigate these experiences, which increase inequality within this already marginalized group. FUNDING: Ministry of Health, Welfare and Sport of the Netherlands and Zorgverzekeraars Nederland (Healthcare Insurers Netherlands).

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11169950/

Record 440

pmcid: PMC10795932

pmid: 37437159

doi: 10.1210/clinem/dgad411

title: Weight Gain After HIV Therapy Initiation: Pathophysiology and Implications.

authors

- Chandiwana NC
- Siedner MJ
- Marconi VC
- Hill A
- Ali MK
- Batterham RL
- Venter WDF

journal: The Journal of clinical endocrinology and metabolism

journalAbbr: J Clin Endocrinol Metab

year: 2024

pubdate: 2024 Jan 18

volume: 109

issue: 2

pages: e478-e487

abstract: Rapid advances in the potency, safety, and availability of modern HIV antiretroviral therapy (ART) have yielded a near-normal life expectancy for most people living with HIV (PLWH). Ironically, considering the history of HIV/AIDS (initially called "slim disease" because of associated weight loss), the latest dilemma faced by many people starting HIV therapy is weight gain and obesity, particularly Black people, women, and those who commenced treatment with advanced immunodeficiency. We review the pathophysiology and implications of weight gain among PLWH on ART and discuss why this phenomenon was recognized only recently, despite the availability of effective therapy for nearly 30 years. We comprehensively explore the theories of the causes, from initial speculation that weight gain was simply a return to health for people recovering from wasting to comparative effects of newer regimens vs prior toxic agents, to direct effects of agents on mitochondrial function. We then discuss the implications of weight gain on modern ART, particularly concomitant effects on

lipids, glucose metabolism, and inflammatory markers. Finally, we discuss intervention options for PLWH and obesity, from the limitations of switching ART regimens or specific agents within regimens, weight-gain mitigation strategies, and potential hope in access to emerging antiobesity agents, which are yet to be evaluated in this population.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10795932/

Record 441

pmcid: PMC11354778

pmid: 39201720

doi: 10.3390/ijms25169034

title: Wilson's Disease-Crossroads of Genetics, Inflammation and Immunity/Autoimmunity: Clinical and Molecular Issues.

authors

- Gromadzka G
- Czerwińska J
- Krzemińska E
- Przybyłkowski A
- Litwin T

journal: International journal of molecular sciences

journalAbbr: Int J Mol Sci

year: 2024

pubdate: 2024 Aug 20

volume: 25

issue: 16

pages:

abstract: Wilson's disease (WD) is a rare, autosomal recessive disorder of copper metabolism caused by pathogenic mutations in the ATP7B gene. Cellular copper overload is associated with impaired iron metabolism. Oxidative stress, cuproptosis, and ferroptosis are involved in cell death in WD. The clinical picture of WD is variable. Hepatic/neuropsychiatric/other symptoms may manifest in childhood/adulthood and even old age. It has been shown that phenotypic variability may be determined by the type of ATP7B genetic variants as well as the influence of various genetic/epigenetic, environmental, and lifestyle modifiers. In 1976, immunological abnormalities were first described in patients with WD. These included an increase in IgG and IgM levels and a decrease in the percentage of T lymphocytes, as well as a weakening of their bactericidal effect. Over the following years, it was shown that there is a bidirectional relationship between copper and inflammation. Changes in serum cytokine concentrations and the relationship between cytokine gene variants and the clinical course of the disease have been described in WD patients, as well as in animal models of this disease. Data have also been published on the occurrence of antinuclear antibodies (ANAs), antineutrophil cytoplasmic antibodies (ANCAs), anti-muscle-specific tyrosine kinase antibodies, and anti-acetylcholine receptor antibodies, as well as various autoimmune diseases, including systemic lupus erythematosus (SLE), myasthenic syndrome, ulcerative colitis, multiple sclerosis (MS), polyarthritis, and psoriasis after treatment with d-penicillamine (DPA). The occurrence of autoantibodies was also described, the presence of which was not related to the type of treatment or the form of the disease (hepatic vs. neuropsychiatric). The mechanisms responsible for the occurrence of autoantibodies in patients with WD are not known. It has also not been clarified whether they have clinical significance. In some patients, WD was differentiated or coexisted with an autoimmune disease, including autoimmune hepatitis or multiple sclerosis. Various molecular mechanisms may be responsible for immunological abnormalities and/or the inflammatory processes in WD. Their better understanding may be important for explaining the reasons for the diversity of symptoms and the varied course and response to therapy, as well as for the development of new treatment regimens for WD.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11354778/

Record 442

pmcid: PMC10230602

pmid: 37266393

doi: 10.1177/26323524231176829

title: 'Out of the frying pan into the fire': a qualitative study of the impact on masculinity for men living with advanced prostate cancer.

authors

- Salifu Y
- Almack K
- Caswell G

journal: Palliative care and social practice

journalAbbr: Palliat Care Soc Pract

year: 2023

pubdate: 2023

volume: 17

issue:

pages: 26323524231176829

abstract: BACKGROUND: Studies have highlighted how advanced prostate cancer causes biographical disruption and presents challenges to masculine identities for men. This article draws on a wider study that focused on the experiences of men living with advanced prostate cancer and their caregivers. Although men's experience of advanced illness is not overlooked in the literature, only a small body of work has taken an in-depth look at men's experiences with advanced prostate cancer and their caregivers in a non-Westernised cultural and social context. OBJECTIVE: To explore how advanced prostate cancer impacts on men's masculine identity from the perspective of patients and their caregivers. METHODS: A qualitative study of men living with advanced prostate cancer (n = 23) and family caregivers (n = 23) in Ghana. We used the Consolidated Criteria for Reporting Qualitative Studies (COREQ) as the reporting guideline. RESULTS: The findings from this study highlight profound challenges for most men to their masculine identities, from both the treatment and the symptoms of advanced prostate cancer within a non-Westernised, patriarchal society. Four main themes were developed. These were the impact on masculinity in terms of: (1) physical changes, (2) sexual ability, (3) socio-economic roles and (4) expressing emotions. Changes in physical appearance, feeling belittled, having no active sexual life and the inability to continue acting as provider and protector of the family made some men describe their situation as one of moving out of the 'frying pan into the fire'. CONCLUSION: This study revealed the impact of advanced prostate cancer on masculine identity. These narratives add a new dimension to what is already known about the impacts on men's masculine identities when dealing with advanced prostate cancer. This knowledge can help improve the care provided to men with advanced prostate cancer with emphasis on the cultures, beliefs and aspirations of these men and their caregivers.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10230602/

Record 443

pmcid: PMC10398431

pmid: 37545925

doi: 10.4314/ahs.v23i1.48

title: A 10-year clinicopathological analysis of ovarian lesions in a tertiary hospital in Southern Nigeria.

authors

- Abasiattai AM
- Nwafor CC
- Utuk NM

journal: African health sciences

journalAbbr: Afr Health Sci

year: 2023

pubdate: 2023 Mar

volume: 23

issue: 1

pages: 459-468

abstract: BACKGROUND: Ovarian tumours are the most lethal of all gynaecological cancers and they are usually diagnosed in advanced stages when the prognosis is very poor. OBJECTIVE: To determine the pattern of ovarian lesions, their frequency, presentation, and associated clinical symptoms in Uyo, Nigeria. METHODS: A 10-year retrospective study of all ovarian specimens that were surgically removed and histologically diagnosed. RESULTS: The patients were between the ages of 5 and 73 years with median age of 34.1 years. Benign tumours occurred most commonly among the 20-39-year age group (31.3%) while malignant tumours were predominant among those aged 50-69 years (10.0%). Surface epithelial tumours (45.4%) were the most common neoplastic tumours while the mature cystic teratoma (33.2%) was the most common tumour overall. Surface epithelial malignancies accounted for 70.6% of all ovarian malignancies and the serous cyst adenocarcinoma (10.2%) was the most common surface epithelial tumour as well as the most common malignant tumour. CONCLUSION: There has been an increase in the number of malignant ovarian specimens in our centre. Though surface epithelial tumours were the most common category of ovarian tumours, overall, the mature cystic teratoma was the most common tumour. Serous cyst adenocarcinoma was the most common surface epithelial tumour and the most common malignant tumour.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10398431/

Record 444

pmcid: PMC10568573

pmid: 37842143

doi: 10.1297/cpe.2023-0032

title: A case of 46,XY complete gonadal dysgenesis with a novel missense variant in SRY.

authors

- Narita C
- Takubo N
- Sammori M
- Matsumura Y
- Shimura K
- Ozaki R
- Haruna H
- Narumi S
- Ishii T
- Hasegawa T
- Shimizu T

journal: Clinical pediatric endocrinology : case reports and clinical investigations : official journal of the Japanese Society for Pediatric Endocrinology

journalAbbr: Clin Pediatr Endocrinol

year: 2023

pubdate: 2023

volume: 32

issue: 4

pages: 235-238

abstract: Disorders of sex development (DSD) with mild external genital abnormalities may be diagnosed after puberty. Here, we report a case of 46,XY complete gonadal dysgenesis with a novel missense variant in sex-determining region Y (SRY) , diagnosed after primary amenorrhea. A 15-yr-old patient presented to our gynecology department with a chief complaint of amenorrhea. The patient was diagnosed with a 46,XY karyotype, and SRY gene positivity. Gonadotropin levels were high, whereas testosterone levels were low. A pelvic magnetic resonance imaging (MRI) revealed a hypoplastic uterus; however, no gonads could be identified. Laparoscopy revealed bilateral streak gonads, fallopian tube-like structures, and the uterus. The gonads were removed based on the risk of gonadal malignancy. Comprehensive genetic analysis of DSD revealed a previously unreported SRY variant, c.271A>T, p.Ser91Cys, and in silico analysis predicted the variant to be pathogenic. The patient was diagnosed with 46,XY complete gonadal dysgenesis with a novel missense variant in SRY . The patient continued female hormone replacement therapy and experienced breast enlargement and cyclic menstruation. Determining the etiology of DSD can be difficult, causing anxiety in patients and their families. In addition to surgical scrutiny, genetic analysis is important to aid in diagnosis and reassure patients and their families.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10568573/

Record 445

pmcid: PMC10497332

pmid: 37705810

doi: 10.1055/s-0043-1770963

title: A New Clinical Classification for Gynecomastia Management and Predictive Outcome.

authors

- Punia S
- Gupta A

journal: Indian journal of plastic surgery : official publication of the Association of Plastic Surgeons of India

journalAbbr: Indian J Plast Surg

year: 2023

pubdate: 2023 Aug

volume: 56

issue: 4

pages: 332-337

abstract: Aim and objectives The aim of this study is to propose a new clinical classification for gynecomastia management based on our experience. Introduction We believe that the prevalent gynecomastia classifications need to be more detailed. Several key aspects that we noticed have either been incompletely represented or not mentioned at all, leaving them unclassified. Materials and Methods This study was conducted at the authors' center over 24 months from May 2020 to May 2022, and included 1,571 patients who were classified and treated as per our new classifications and small iterations were done to strengthen the classification. Results We were able to better classify our patients and previously unclassified situations. Skin sagging or a potential for skin sagging and puffy nipples were managed better. This classification allowed recommendation of each step of the procedure, resulting in high patient satisfaction, as all problem areas were sorted and outcome that was

explained to them preoperatively was clearly visible postoperatively. Conclusion The new clinical classification for gynecomastia is a simple, clinical, straightforward, comprehensive, grading system, the use of which has helped the authors and the team in achieving precise and predictable results while ensuring patient satisfaction.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10497332/

Record 446

pmcid: PMC10356841

pmid: 37468498

doi: 10.1038/s41467-023-40064-9

title: A preclinical secondary pharmacology resource illuminates target-adverse drug reaction associations of marketed drugs.

authors

- Sutherland JJ
- Yonchev D
- Fekete A
- Urban L

journal: Nature communications

journalAbbr: Nat Commun

year: 2023

pubdate: 2023 Jul 19

volume: 14

issue: 1

pages: 4323

abstract: In vitro secondary pharmacology assays are an important tool for predicting clinical adverse drug reactions (ADRs) of investigational drugs. We created the Secondary Pharmacology Database (SPD) by testing 1958 drugs using 200 assays to validate target-ADR associations. Compared to public and subscription resources, 95% of all and 36% of active (AC50 < 1 μM) results are unique to SPD, with bias towards higher activity in public resources. Annotating drugs with free maximal plasma concentrations, we find 684 physiologically relevant unpublished off-target activities. Furthermore, 64% of putative ADRs linked to target activity in key literature reviews are not statistically significant in SPD. Systematic analysis of all target-ADR pairs identifies several putative associations supported by publications. Finally, candidate mechanisms for known ADRs are proposed based on SPD off-target activities. Here we present a freely-available resource for benchmarking ADR predictions, explaining phenotypic activity and investigating clinical properties of marketed drugs.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10356841/

Record 447

pmcid: PMC10579622

pmid: 37854731

doi: 10.7759/cureus.45383

title: A Prospective Study of the Age at Menarche in North Indian Girls, Its Association With the Tanner Stage, and the Secular Trend.

authors

- Bajpai A
- Bansal U

- Rathoria R
- Rathoria E
- Singh V
- Singh GK
- Ahuja R

journal: Cureus

journalAbbr: Cureus

year: 2023

pubdate: 2023 Sep

volume: 15

issue: 9

pages: e45383

abstract: Background Menarche is an important event in a female's reproductive health. However, the age at menarche is variable and has implications on the future health of the girl. The purpose of this study was to estimate the mean age at menarche of North Indian adolescent girls and its association with the Tanner stage and to study the trend of menarcheal age in India in the 21st century. Materials & methods A longitudinal descriptive study was conducted on 470 healthy girls aged nine to 16 years in expectant menarche (Tanner stage II). They were followed for six months to check for the attainment of menarche. Those who achieved menarche were grouped in Group I and the rest in Group II. The data were analyzed using the independent t-test. Results Menarche was achieved by 263 girls in the six-month follow-up period. The mean (SD) age of menarche was 13.13 (1.23) years. Group I girls were mostly in Tanner stage IV and above. Group II girls had a mean (SD) age of 11.53 (1.1) years and were mostly in Tanner stage III or below. The estimated decline in the age of menarche in the 21st century was about 0.41 years per decade. Conclusion The girls who achieved menarche had a significantly higher age and Tanner stage of sexual maturity than girls who did not achieve menarche in the study period. Tanner stage is a better measure to estimate the pubertal onset than age. The studies on the age of menarche in the current century reveal that the declining trend in the menarcheal age is continuing in India.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10579622/

Record 448

pmcid: PMC10084942

pmid: 37050986

doi: 10.7759/cureus.36019

title: A Rare Case of Primary Breast Diffuse Large B-cell Lymphoma in an Acquired Immunodeficiency Syndrome Patient.

authors

- Lockwood MA
- Noginskiy I
- Kalavar M

journal: Cureus

journalAbbr: Cureus

year: 2023

pubdate: 2023 Mar

volume: 15

issue: 3

pages: e36019

abstract: Primary breast lymphoma (PBL) is not a commonly seen subtype of breast cancer, and it is also unusual for an extranodal variant of diffuse large B-cell lymphomas (DLBCLs) to appear in the breast. In this case report, we recount the presentation of painful masses in the right axillary and right breast regions in an acquired immunodeficiency syndrome (AIDS) patient, shortly after a mammogram described her breast lesion as BI-RADS 3, probably benign, in the breast imaging reporting and data system. This case demonstrated that painful breast and axillary masses in an acquired immunodeficiency syndrome (AIDS) patient can grow quickly, be misdiagnosed, and require an expedient workup, as extranodal DLBCL can be a debilitating disease.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10084942/

Record 449

pmcid: PMC10256625

pmid: 37305344

doi: 10.1093/jscr/rjad338

title: A rare case: synchronous ipsilateral breast implant-associated anaplastic large cell lymphoma and invasive ductal carcinoma.

authors

- Safari A
- Hamza S
- Paton D

journal: Journal of surgical case reports

journalAbbr: J Surg Case Rep

year: 2023

pubdate: 2023 Jun

volume: 2023

issue: 6

pages: rjad338

abstract: A 78-year-old female with a history of cosmetic breast implants presented with unilateral breast enlargement and was subsequently diagnosed with stage IA breast implant-associated anaplastic large cell lymphoma (BIA-ALCL) as well as stage IB ipsilateral synchronous invasive ductal carcinoma (IDC). Her assessment included bilateral breast ultrasounds,mammograms and MRIs with right-sided fine needle aspiration of peri-implant fluid, core biopsy of right breast mass and a whole-body positron emission tomography scan. She was surgically treated with bilateral capsulectomy, implant removal and mastectomy. No adjuvant treatment was required for the BIA-ALCL. The IDC required adjuvant chemotherapy, radiotherapy and endocrine therapy. This rare case highlights the paramount importance of thorough evaluation of suspected BIA-ALCL patients for synchronous breast pathologies. We conclude with a concise summary of the salient points on evaluation and management of BIA-ALCL for surgeons.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10256625/

Record 450

pmcid: PMC10513005

pmid: 37780976

doi: 10.1259/bjrcr.20220160

title: A rarely described Kimura's disease of the breast.

authors

- Almohawes EA
- Zakareia Y
- Dumiati ZA
- Zaki WA
- Alfudhili KM

journal: BJR case reports

journalAbbr: BJR Case Rep

year: 2023

pubdate: 2023 Oct

volume: 9

issue: 5

pages: 20220160

abstract: Kimura's disease is a rare chronic inflammatory disease of unknown aetiology. The majority of cases are reported in middle-aged Asian males and rarely seen in other ethnicities. Patients classically present with chronic single or multiple painless subcutaneous masses, lymphadenopathies, hypereosinophilia and elevated serum immunoglobulin E. The disease usually affects the head and neck area, however, rare involvement of other locations such as axilla, upper limbs, popliteal region and abdomen has been reported. Here, we report a rarely encountered Kimura's disease of the breast and axillary lymph nodes in a middle-aged female. In this report, we will describe the main clinical, radiological and pathological features to raise the awareness about this indolent disease.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10513005/

Record 451

pmcid: PMC10010038

pmid: 36915127

doi: 10.1186/s12902-023-01310-9

title: A retrospective cohort study of tamoxifen versus surgical treatment for ER-positive gynecomastia.

authors

- He W
- Wei W
- Zhang Q
- Lv R
- Qu S
- Huang X
- Ma J
- Zhang P
- Zhai H
- Wang N

journal: BMC endocrine disorders

journalAbbr: BMC Endocr Disord

year: 2023

pubdate: 2023 Mar 13

volume: 23

issue: 1

pages: 62

abstract: BACKGROUND: Gynecomastia is a common condition in clinical practice. The present study aimed to review the clinical data of ER-positive gynecomastia patients treated by tamoxifen (TAM) versus surgery and discussed the clinical effects of the two treatment strategies. METHOD: We retrospectively collected the clinical indicators of patients with unilateral or bilateral gynecomastia who received treatment at our hospital between April 2018 and December 2021. Depending on the treatment received, the patients were divided into TAM and surgery groups. RESULT: A total of 170 patients were recruited, including 91 patients in TAM group and 79 patients in surgery group. The age of the patients differed significantly between the TAM and surgery groups ($P < 0.01$). The estrogen level was closer in patients with stable and progressive disease, but significantly different in patients of glandular shrinkage in TAM group ($P < 0.01$). The proportion of patients achieving stable disease was higher among those with clinical grade 1-2. Among patients classified as clinical grade 3, the proportion of patients achieving glandular shrinkage of the breast was higher after TAM treatment ($P < 0.05$). The age and length of hospital stay were significantly different in patients undergoing open surgery than minimally invasive rotary cutting surgery and mammoscopic-assisted glandular resection ($P < 0.01$). Patients had significantly different complications including mild postoperative pain, hematoma, nipple necrosis, nipple paresthesias and effusions among the surgery subgroups (all $P < 0.05$). The estrogen level and the type of surgery were significantly different between the surgical recurrence and non-recurrence subgroups ($P < 0.05$). The difference in the thickness of glandular tissues upon the color Doppler ultrasound also reached a statistical significance between the two groups ($P = 0.050$). An elevated estrogen level was a factor leading to TAM failure. Among surgical patients, the thickness of glandular tissues, estrogen level, and type of surgery performed were risk factors for postoperative recurrence (all $P < 0.05$). CONCLUSION: Both treatment strategies can effectively treat gynecomastia, but different treatment methods can benefit different patients. TAM treatment is more beneficial than surgery for patients who cannot tolerate surgery, have a low estrogen level, and are clinical grade 1-2. Surgery treatment is better than TAM for patients of clinical grade 3. Different surgery options may lead to different complications. Patients with a greater glandular tissue thickness and a higher estrogen level were shown to have a higher risk of recurrence.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10010038/

Record 452

pmcid: PMC10451580

pmid: 37636187

doi: 10.4103/jfmpc.jfmpc_1566_22

title: A single group trial (before - after) on impact of physiotherapeutic intervention package on relief from low back pain of pregnant women.

authors

- Sarkar PK
- Kumar A
- Singh A
- Sharma S
- Bhattacharya S

journal: Journal of family medicine and primary care

journalAbbr: J Family Med Prim Care

year: 2023

pubdate: 2023 Jun

volume: 12

issue: 6

pages: 1078-1082

abstract: INTRODUCTION: More than half of the women have low back pain (LBP) at some point during pregnancy; the intensity of back pain increases as the pregnancy progresses. Such changes increase their risk for musculoskeletal disorders and fall injuries. AIM AND OBJECTIVE: Our study's objective was to estimate the prevalence of LBP in antenatal and postnatal cases of pregnancy in PGIMER, Chandigarh and to ascertain impact of a training intervention on incidence and severity of LBP in the study subjects. METHODOLOGY: A single group trial (before and after) on impact of physiotherapeutic intervention package on relief from LBP in pregnant women in between 20 and 35 years of age group were selected. The study duration was one year. The numeric pain rating scale was taken for measuring pain intensity at different levels. Where 0 indicates no pain, 1-3 indicates mild pain (which may be nagging, annoying, and interfering little with activities of daily livings (ADLs)), 4-6 indicates moderate pain (interferes significant with ADLs), and 7-10 indicates severe pain (Disabling, unable to perform ADLs). As per available time, it was decided to include at least 30 subjects in the study. RESULTS: Initially 45 patients were enrolled. Of those, 30 patients were suffering from backache, that is, 67% of the total and 33% did not have any backache. There were no backaches reported in the first trimester, five cases were in the second trimester, and 18 cases were in the third trimester. Of 30 patients, 11 were less than 25 years of age and remaining 19 patients were in the age group of 26-35 years.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10451580/

Record 453

pmcid: PMC10526244

pmid: 37760485

doi: 10.3390/cancers15184516

title: A Systematic Review of Breast Implant-Associated Squamous Cell Carcinoma.

authors

- Niraula S
- Katel A
- Barua A
- Weiss A
- Strawderman MS
- Zhang H
- Manrique O
- O'Connell A
- Pandey SR
- Dhakal A

journal: Cancers

journalAbbr: Cancers (Basel)

year: 2023

pubdate: 2023 Sep 12

volume: 15

issue: 18

pages:

abstract: Breast augmentation is considered safe, but rare cases of breast implant-associated squamous cell carcinoma (BIA-SCC) have been reported. This study aimed to systematically review published cases of BIA-SCC, providing valuable clinical data. The review included 14 articles and 18 cases of BIA-SCC. An increasing trend in reported BIA-SCC cases was observed, with four cases in the 1990s and 14 cases since 2010. The mean age of affected patients was 56 years, and symptoms typically appeared around 21 years after breast augmentation. Silicone implants used in cosmetic procedures were most commonly associated with BIA-SCC.

Implant removal was necessary in all cases, and some patients required a mastectomy. Treatment approaches varied, with the selective use of chemotherapy and/or radiotherapy. The estimated 6-month mortality rate was 11.1%, while the 12-month mortality rate was 23.8%. The estimated 6-month mortality rate should be cautiously interpreted due to the limited sample size. It appears lower than the rate reported by the American Society of Plastic Surgeons, without clear reasons for this discrepancy. This study highlights the importance of enhanced monitoring and information sharing to improve detection and management of BIA-SCC. Healthcare providers should maintain vigilance during the long-term follow-up of breast augmentation patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10526244/

Record 454

pmcid: PMC10248091

pmid: 37305071

doi: 10.1016/j.adro.2023.101263

title: Accelerated Partial Breast Irradiation: Association of Dosimetric Parameters With Patient-Reported Outcomes.

authors

- Lapen K
- Fiasconaro M
- Zhang Z
- Abaya A
- McCormick B
- Xu AJ
- Cuaron J
- Powell SN
- Kahn AJ
- Gillespie EF
- Braunstein LZ

journal: Advances in radiation oncology

journalAbbr: Adv Radiat Oncol

year: 2023

pubdate: 2023 Nov-Dec

volume: 8

issue: 6

pages: 101263

abstract: PURPOSE: Accelerated partial breast irradiation (APBI) after breast-conserving surgery offers a well-tolerated adjuvant radiation therapy option for patients with breast cancer. We sought to describe patient-reported acute toxicity as a function of salient dosimetric parameters during and after an APBI regimen of 40 Gy in 10 once-daily fractions. METHODS AND MATERIALS: From June 2019 to July 2020, patients undergoing APBI were assigned a weekly, response-adapted, patient reported outcomes-common terminology criteria for adverse events-based acute toxicity assessment. Patients reported acute toxicity during treatment and for up to 8 weeks after treatment. Dosimetric treatment parameters were collected. Descriptive statistics and univariable analyses were used to summarize patient-reported outcomes and their correlation to corresponding dosimetric measures, respectively. RESULTS: Overall, 55 patients who received APBI completed a total of 351 assessments. Median planning target volume was 210 cc (range, 64-580 cc), and median planning target volume:ipsilateral breast volume ratio was 0.17 (range, 0.05-0.44). Overall, 22% of patients reported moderate breast enlargement and 27% reported maximum skin toxicity as severe or very severe. Furthermore, 35% of

patients reported fatigue, and 44% of patients reported pain in the radiated area as moderate to very severe. Median time to first report of any moderate to very severe symptom was 10 days (interquartile range, 6-27 days). By 8 weeks after APBI, most patients reported resolution of symptoms, with 16% reporting residual moderate symptoms. Upon univariable analysis, none of the ascertained salient dosimetric parameters were associated with maximum symptoms or with the presence of moderate to very severe toxicity. CONCLUSIONS: Weekly assessments during and after APBI showed that patients experienced moderate to very severe toxicities, most commonly skin toxicity, but that these typically resolved by 8 weeks after radiation therapy. More comprehensive evaluations among larger cohorts are warranted to define the precise dosimetric parameters that correspond to outcomes of interest.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10248091/

Record 455

pmcid: PMC10617457

pmid: 37915344

doi: 10.1177/22925503211051120

title: Activin A and Follistatin Serum Concentrations in Breast Augmentation Patients.

authors

- Zeplin PH

journal: Plastic surgery (Oakville, Ont.)

journalAbbr: Plast Surg (Oakv)

year: 2023

pubdate: 2023 Nov

volume: 31

issue: 4

pages: 377-382

abstract: Background: Capsular contracture is caused by an excessive fibrotic reaction similar as observed in other progressive fibrotic disorders. For their pathogenesis, several studies confirmed the importance of activins and follistatin. The aim of this study was to determine and analyze serum levels of Activin A and follistatin in patients with capsular contracture after aesthetic breast augmentation. Methods: The study included 361 female patients who underwent primary aesthetic breast augmentation, came for control examination after breast augmentation or for revision operation because of capsular contracture. Blood samples were taken and using a specific ELISA to determine the serum concentration levels of Activin A and Follistatin. Results: Ninety-six patients (n = 96), who developed a capsular contracture Baker ≥°III and underwent revision surgery were collected (capsular fibrosis group). One-hundred and fourteen patients (n = 114) were asymptomatic for capsular fibrosis Baker ≥°III after primary breast augmentation and 33 (n = 33) of them had developed no capsular fibrosis after more than 10 years (long-term group). For control group, blood samples were taken from 167 patients (n = 167) before primary aesthetic breast augmentation. Serum Activin A levels were significantly higher in the long-term Group compared with those in the capsular fibrosis- and the control groups. Follistatin levels were significantly lower in the capsular fibrosis group compared to the control- and the long-term groups. A small amount of control group patients (n = 16) developed a capsular fibrosis within 2 years after primary breast augmentation with significant lower follistatin levels. Retrospectively, they showed significantly lower serum follistatin levels than the control group even before the onset of capsular contracture. Conclusions: Capsular fibrosis has no effect on Activin A serum levels. In contrast, follistatin serum levels are lower in patients with capsular fibrosis. These results show that besides many other factors, a dysregulation of the Activin-follistatin axis may have importance on the pathogenesis of capsular contracture.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10617457/

Record 456

pmcid: PMC10129272

pmid: 37113825

doi: 10.1097/MS9.0000000000000447

title: Adrenocortical carcinoma in a 10-month-old infant: A literature review and a rare case report.

authors

- Al-Ghotani B
- Alabdallah E
- Shaaban V
- Nemer F
- Alsneeh A
- Sharif FA
- Dalati H
- Mansour M

journal: Annals of medicine and surgery (2012)

journalAbbr: Ann Med Surg (Lond)

year: 2023

pubdate: 2023 Apr

volume: 85

issue: 4

pages: 1197-1205

abstract: UNLABELLED: Adrenocortical carcinoma (ACC) in children is a rare condition. The annual incidence of ACC is extremely low, with only 0.2-0.3 cases per million children. The clinical presentations of ACC are numerous, such as terminal hair appearance, pubertal progress, hypercortisolism, enlarged clitoris, acne, systemic arterial hypertension, weight gain, and voice change. CASE PRESENTATION: A 10-month-old female infant presented by her parents to the Department of Endocrinology with a mass on the right adrenal gland and Cushing's syndrome symptoms. Surgery was performed. The death occurred after two times resuscitation due to sudden cardiac arrest. CLINICAL DISCUSSION: The adrenal gland consists of two distinct parts. Different types of tumors arise from each part of the adrenal gland. The most common tumor in adrenomedullary tumors was neuroblastoma which accounted for 60.4% of adrenal tumors. ACC in children is a rare condition. The etiology of ACTs is unclear. CONCLUSION: This case emphasizes that early diagnosis has a considerable role in preventing major complications. Also to advise considering ACC as a differential diagnosis when similar symptoms are found in an infant.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10129272/

Record 457

pmcid: PMC10204266

pmid: 37228316

doi: 10.1093/asjof/ojad021

title: Aesthetic Refinements in Male Chest Lifting.

authors

- Austin RE
- Milkovich J
- Lista F
- Ahmad J

journal: Aesthetic surgery journal. Open forum

journalAbbr: Aesthet Surg J Open Forum

year: 2023

pubdate: 2023

volume: 5

issue:

pages: ojad021

abstract: The authors describe aesthetic refinements to the approach for male chest lifting in male patients with grade 3 gynecomastia and/or significant chest skin excess. An inferior pedicle is used to transpose the nipple-areolar complex allowing preservation of pigment and sensation, liposuction and direct excision are used to reduce volume and excess skin, and the resulting curvilinear scar along the inferior and lateral border of the chest provide a more masculine appearance. Early experience with this technique has shown it to be safe and effective. Perioperative management and the detailed steps of the procedure are outlined.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10204266/

Record 458

pmcid: PMC10578417

pmid: 37908278

doi: 10.1210/jcemcr/luac033

title: An Interesting Case of Hypogonadism: Workup in a Phenotypic Male Reveals XX Genotype.

authors

- Sharma T
- Mehta A

journal: JCEM case reports

journalAbbr: JCEM Case Rep

year: 2023

pubdate: 2023 Jan

volume: 1

issue: 1

pages: luac033

abstract: We present a patient case referred for evaluation of male hypogonadism with gynecomastia. On examination, he was noted to have microtestis, shorter than expected height, and bilateral gynecomastia. Further investigation revealed XX genotype and on fluorescence in situ hybridization analysis confirmed the SRY gene was present on the short arm of 1 X chromosome. This case highlights the importance of detailed history and examination and the indication for genetic counseling in selected cases.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10578417/

Record 459

pmcid: PMC10663875

pmid: 38021710

doi: 10.7759/cureus.47502

title: Anaesthetic Management for Mastectomy in a Male With Unilateral Gynecomastia: The Utilization of Thoracic Segmental Spinal Anaesthesia and Erector Spinae Plane Block.

authors

- Paul A
- Borkar A

journal: Cureus

journalAbbr: Cureus

year: 2023

pubdate: 2023 Oct

volume: 15

issue: 10

pages: e47502

abstract: Males are frequently affected by gynecomastia, a benign proliferative glandular tissue condition of the breast. Gynecomastia is usually treated with surgery to remove breast tissue. Using erector spinae plane block and thoracic segmental spinal anaesthesia in place of typical general anaesthesia during breast procedures has become more common in recent years. This case report presents the management of a 24-year-old male with long-standing left breast gynecomastia. Using a combination of erector spinae plane block and thoracic segmental spinal anaesthesia, the patient had the breast tissue excised. The regulation of the neuroendocrine stress response, lower need for analgesics after surgery, and decreased postoperative nausea and vomiting are among the many benefits of the anaesthetic methods. With better patient outcomes, fewer surgical complications, and efficient postoperative pain management, these methods offer a compelling substitute for general anaesthesia. The range of surgical scenarios in which these techniques can be applied could be expanded by additional research and clinical experience.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10663875/

Record 460

pmcid: PMC9902018

pmid: 36749226

doi: 10.1097/MD.0000000000032753

title: Anaesthetic management of cerebral arteriovenous malformation hemorrhage during pregnancy: A case series.

authors

- Ji Y
- Liang Y
- Liu B
- Wang Y
- Li L
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- Dong N
- Xiong W
- Yue H
- Jin X

journal: Medicine

journalAbbr: Medicine (Baltimore)

year: 2023

pubdate: 2023 Feb 3

volume: 102

issue: 5

pages: e32753

abstract: RATIONALE: There is no clear consensus guidance for anesthesiologists on how to manage patients with cerebral arteriovenous malformation (cAVM) rupture and hemorrhage during pregnancy who need craniotomy. Our objective was to review the anesthesia management of pregnant women who underwent resection of cAVM at our institution and to provide opinions and suggestions. PATIENT CONCERNS: Herein, we report of 3 patients with cAVM rupture and hemorrhage during pregnancy who underwent neurosurgery at the 22nd, 28th, and 20th weeks of pregnancy. DIAGNOSES: All 3 patients were admitted to the emergency department of our hospital due to sudden symptoms. Subsequently, their head imaging results confirmed the rupture and hemorrhage of cAVM. The rupture and hemorrhage of cAVM during pregnancy has a low incidence and high mortality, which seriously endangers the safety of the mother and fetus. For this emergency condition, craniotomy for removing intracranial lesions and clear hematoma can result in a chance of a successful delivery. Especially in the second and third trimesters of pregnancy, the management goal of anesthesia is to ensure the maternofetal safety and to maintain continuous pregnancy. INTERVENTIONS: This article describes the process of intraoperative anesthesia management and maternal-fetal outcomes and discusses the key issues for the anesthesia management of cAVM rupture during pregnancy, including considerations of physiological changes during pregnancy and anesthesia medication, intraoperative monitoring, the maintenance of hemodynamic stability, and the control of intracranial pressure, among other considerations. Resection of intracranial lesions should be performed whenever possible while maintaining the pregnancy for better maternal and infant outcomes. OUTCOMES: The operations of the 3 pregnant women were successfully completed under our detailed anesthesia planning and careful anesthesia management. All the patients recovered well after the operation, and underwent cesarean section to give birth smoothly. LESSONS: The preservation of pregnancy under cAVM resection is a complex challenge for anesthesiologists, and these 3 cases provide an extensive amount of experience for anesthesia management in similar situations. Detailed anesthesia planning and careful anesthesia management by anesthesiologists are important guarantees for good maternal and fetal outcomes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9902018/

Record 461

pmcid: PMC10071202

pmid: 37002862

doi: 10.1177/15579883231165626

title: Analysis of Knowledge About Male Breast Cancer Among Patients at Tertiary Medical Center.

authors

- Altiner S
- Altiner ÖT
- Büyükkasap Ç
- Uğraş Dikmen A
- Pekcici MR
- Erel S

journal: American journal of men's health

journalAbbr: Am J Mens Health

year: 2023

pubdate: 2023 Mar-Apr

volume: 17

issue: 2

pages: 15579883231165626

abstract: In the overall population, the incidence of breast cancer in men is lower than in women. Men's breast cancer awareness is affected both by the low incidence of breast cancer in men and by the presence of a perception that breast cancer can only be seen in women in society. This study aims to determine this awareness and guide future studies on improving social awareness. This study examined male and female patients aged 18 to 75 years who were admitted to our hospital's general surgery outpatient clinic. A questionnaire containing questions about male breast cancer was administered to the patients, and the study was conducted face-to-face voluntarily. A total of 411 patients, 270 female and 141 male, participated in the study. The results showed that 61.1% of the participants were unaware of the possibility of breast cancer in men. Evaluation of the relationship between awareness and gender revealed that women were more knowledgeable than men (p = .006). Educational status also had a significant influence on awareness (p = .001). Awareness of male breast cancer in society is low. Raising public awareness of this issue will enable men to be diagnosed earlier, at a lower stage, and thus to better respond to treatment, increasing their survival time.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10071202/

Record 462

pmcid: PMC10494349

pmid: 37697362

doi: 10.1186/s12887-023-04265-x

title: Analysis of risk factors of precocious puberty in children.

authors

- Dong Y
- Dai L
- Dong Y
- Wang N
- Zhang J
- Liu C
- Li Z
- Chu L
- Chen S

journal: BMC pediatrics

journalAbbr: BMC Pediatr

year: 2023

pubdate: 2023 Sep 11

volume: 23

issue: 1

pages: 456

abstract: BACKGROUND: The purpose of this study is to explore the related factors of precocious puberty in children. METHODS: 1239 children who underwent physical examination in our hospital from January 2020 to December 2022 were analyzed, including 198 precocious children and 1041 normal children. According to the age of 198 precocious children and 1041 normal children, 205 normal children were selected, and the remaining 836 normal children were excluded. They were divided into precocious group and normal group. The general data of the two groups were recorded. Logistic regression was used to analyze the influencing factors of precocious puberty in children. RESULTS: There were statistically significant differences (P < 0.05) between the two groups in sex, bone age, daily exercise time, E2, FSH, LH, leptin, mother's menarche time, living environment, consumption of nutritional supplements, consumption of foods containing pigments and

preservatives, consumption of high-protein foods, and sleeping time. The multifactor logistic regression analysis shows that the risk factors of children's precocious puberty included gender (female), bone age (> 10 years old), and daily exercise time (< 0.9 h), E2 (≥ 66.00pmol/L), FSH (≥ 6.00U/L), LH (≥ 3.50U/L), leptin (≥ 8.00 μ G/L), mother's menarche time (< 12 years old), living environment (chemical industry zone), consumption of nutritional supplements (often), consumption of high-protein food (often), and sleep time (< 10 h). CONCLUSION: In conclusion, children's gender, bone age, exercise habits, E2, FSH, LH, leptin, mother's menarche time, living environment, eating habits, sleep time and other factors are closely related to precocious puberty in children. Reminding parents to actively prevent related factors in clinical work is helpful to prevent the occurrence of precocious puberty in children.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10494349/

Record 463

pmcid: PMC10080485

pmid: 37035395

doi: 10.21037/tp-23-111

title: Analysis of the efficacy of Dabuyin pill combined with gonadotropin-releasing hormone analogue in the treatment of central precocious puberty girls based on network pharmacology.

authors

- Chen X
- Zheng M
- Fei X
- Ma X

journal: Translational pediatrics

journalAbbr: Transl Pediatr

year: 2023

pubdate: 2023 Mar 31

volume: 12

issue: 3

pages: 364-374

abstract: BACKGROUND: Traditional Chinese medicine (TCM) believes that central precocious puberty (CPP) is affected by the imbalance of kidney Yin and Yang. Dabuyin pill is a recipe for nourishing Yin and lowering fire. The network pharmacology method was used to analyze the active components, action targets, and molecular pathways of Dabuyin pill in the treatment of CPP. METHODS: The main chemical components of Dabuyin pill were obtained from the Integrative database of Traditional Chinese Medicine enhanced by Symptom Mapping (SymMap) database and Traditional Chinese Medicine Systems Pharmacology (TCMSP), and compound targets were retrieved from SymMap and the Encyclopedia of Traditional Chinese Medicine (ETCM). Disease targets were retrieved from the DisGeNET and Gene Expression Omnibus (GEO) databases, and the intersection of compound targets and disease targets was performed to obtain the prediction targets of Dabuyin pill acting on CPP. The key targets enriched by Database for Annotation, Visualization, and Integrated Discovery (DAVID) were then used for Gene Ontology (GO) functional enrichment and Kyoto Encyclopedia of Genes and Genomes (KEGG) pathway analysis. RESULTS: GO analysis showed that the biological functions of Dabuyin pill in the treatment of key targets of CPP mainly involved apoptosis, nitric oxide synthesis, estradiol response, angiogenesis, inflammation, and so on. KEGG pathway analysis was mainly enriched in the tumor necrosis factor (TNF) signaling pathway, phosphatidylinositol-3 kinase/protein kinase B (PI3K/Akt) signaling pathway, hypoxia-inducible factor-1 (HIF-1) signaling pathway, and apoptosis. Among them, the regulation effect of Dabuyin pill prescription on apoptosis may both act on TP53 and different signaling pathways of apoptosis, thus playing a synergistic role. CONCLUSIONS: Dabuyin pill combined with GnRHa for the prevention and treatment of CPP in girls can effectively intervene CPP, and the effect of Dabuyin pill on sex

hormones is one of its protective mechanisms against CPP.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10080485/

Record 464

pmcid: PMC10601667

pmid: 37900552

doi: 10.1159/000531578

title: Arbeitsgemeinschaft Gynäkologische Onkologie Recommendations for the Diagnosis and Treatment of Patients with Early Breast Cancer: Update 2023.

authors

- Park-Simon TW
- Müller V
- Jackisch C
- Albert US
- Banys-Paluchowski M
- Bauerfeind I
- Blohmer JU
- Budach W
- Dall P
- Ditsch N
- Fallenberg EM
- Fasching PA
- Fehm T
- Friedrich M
- Gerber B
- Gluz O
- Harbeck N
- Hartkopf AD
- Heil J
- Huober J
- Kolberg-Liedtke C
- Kreipe HH
- Krug D
- Kühn T
- Kümmel S
- Loibl S
- Lüftner D
- Lux MP
- Maass N
- Mundhenke C
- Reimer T
- Rhiem K
- Rody A

- Schmidt M
- Schneeweiss A
- Schütz F
- Sinn HP
- Solbach C
- Solomayer EF
- Stickeler E
- Thomssen C
- Untch M
- Witzel I
- Wöckel A
- Wuerstlein R
- Janni W
- Thill M

journal: Breast care (Basel, Switzerland)

journalAbbr: Breast Care (Basel)

year: 2023

pubdate: 2023 Aug

volume: 18

issue: 4

pages: 289-305

abstract: BACKGROUND: Each year the interdisciplinary Arbeitsgemeinschaft Gynäkologische Onkologie (AGO), German Gynecological Oncology Group Breast Committee on Diagnosis and Treatment of Breast Cancer provides updated state-of-the-art recommendations for early and metastatic breast cancer. SUMMARY: The updated evidence-based treatment recommendation for early and metastatic breast cancer has been released in March 2023. KEY MESSAGES: This paper concisely captures the updated recommendations for early breast cancer chapter by chapter.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10601667/

Record 465

pmcid: PMC10769481

pmid: 38187077

doi: 10.17925/EE.2023.19.2.9

title: Are Prepubertal Gynaecomastia and Premature Thelarche Linked to Topical Lavender and Tea Tree Oil Use?

authors

- Braunstein EW
- Braunstein GD

journal: TouchREVIEWS in endocrinology

journalAbbr: touchREV Endocrinol

year: 2023

pubdate: 2023 Nov

volume: 19

issue: 2

pages: 60-68

abstract: Various studies, conducted since 2007, have reported a total of eight boys with prepubertal gynaecomastia and four girls with premature thelarche following exposure to lavender and/or tree tea oil. All patients experienced regression of the breast tissue after they stopped using these oils. Both of these essential oils, and several of their constituents, have oestrogenic and antiandrogenic activity in vitro . However, limited dermal penetration of some of the components means that the in vitro findings cannot be extrapolated to the in vivo situation. There are unanswered questions as to how much lavender or tea tree oil was actually present in the skincare products used by the children and a lack of information about exposure to other agents. Furthermore, since both prepubertal gynaecomastia and premature thelarche often spontaneously regress, it cannot be concluded that the use of lavender and/or tree tea oil is the cause of the gynaecomastia and thelarche in these children.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10769481/

Record 466

pmcid: PMC10624627

pmid: 37400541

doi: 10.1038/s41390-023-02706-y

title: Association between infant breastfeeding practices and timing of peak height velocity: A nationwide longitudinal survey in Japan.

authors

- Higuchi Y
- Matsumoto N
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journal: Pediatric research

journalAbbr: Pediatr Res

year: 2023

pubdate: 2023 Nov

volume: 94

issue: 5

pages: 1845-1854

abstract: BACKGROUND: Several studies have discovered an association between infant feeding practices and puberty timing; however, most have involved female cohorts. We investigated the association between infant feeding practices and the timing of peak height velocity in boys and girls. METHODS: Data on infant feeding methods and anthropometric measurements were collected from a nationwide Japanese birth cohort study. The age at peak height velocity (APV, years) was estimated and compared. Subsequently, the effects of breastfeeding duration were analyzed. RESULTS: Of the 13,074 eligible participants, 650, 9455, and 2969 were formula-, mixed-, and exclusively breastfed, respectively. Among girls, the mean APV was significantly later in the mixed-fed (standardized regression coefficient (β): 0.094, 95% confidence interval (CI): 0.004-0.180) and exclusively breastfed (β: 0.150, 95% CI: 0.056-0.250) groups than in the formula-fed group. Among boys, the mean APV was not significantly different among the three groups; however, a sensitivity analysis that excluded

preterm birth revealed more significantly delayed APV in the breastfed-only group compared to the formula-fed group. Furthermore, a multiple linear regression model revealed that a longer breastfeeding period was associated with later APV. CONCLUSIONS: Infant breastfeeding practices can affect the timing of peak height velocity in both boys and girls. IMPACT: Several studies have discovered an association between infant feeding practices and puberty timing; however, most have involved female cohorts. Age at peak height velocity, derived from longitudinal height measurements, is a useful marker of secondary sexual maturity milestones in boys and girls. A Japanese birth cohort study revealed that breastfed children had a later age at peak height velocity than their formula-fed counterparts; this was more prominent among girls than boys. Furthermore, a duration-effect relationship was observed, where longer breastfeeding duration was associated with a later age at peak height velocity.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10624627/

Record 467

pmcid: PMC10327631

pmid: 37424740

doi: 10.25100/cm.v54i1.4625

title: Autoimmune/inflammatory syndrome induced by adjuvants (ASIA), medical treatment of severe systemic compromise: case report.

authors

- Puerta Sarmiento GE
- Modragón I
- Echeverri A
- Sua LF
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- Aguirre-Valencia D

journal: Colombia medica (Cali, Colombia)

journalAbbr: Colomb Med (Cali)

year: 2023

pubdate: 2023 Jan-Mar

volume: 54

issue: 1

pages: e5004625

abstract: CASE DESCRIPTION: A 42-year-old woman with severe pulmonary and mediastinal inflammatory involvement, secondary to infiltration of a silicone-related allogenic material with systemic migration. CLINICAL FINDINGS: The patient developed esophageal and bronchial stenosis, recurrent infections, malnutrition, and respiratory deterioration, making surgical removal of the allogenic material impossible. TREATMENT AND OUTCOME: Clinical and radiological improvement was achieved after treatment with multiple intravenous and oral immunomodulators. CLINICAL RELEVANCE: Autoimmune/inflammatory syndrome induced by adjuvants (ASIA) is a heterogeneous disease resulting from exposure to allogenic substances in a susceptible subject. These substances cause autoimmune or autoinflammatory phenomena. Since ASIA was described ten years ago, its diagnostic criteria are still under discussion, with an uncertain prognosis. The ideal therapy is based on eliminating the causative substance, but this is not always possible. Therefore, it is necessary to start an immunomodulatory treatment, using it in this patient, a scheme that had not been previously reported in the literature.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10327631/

Record 468

pmcid: PMC10711344

pmid: 38090464

doi: 10.7759/cureus.48644

title: Beyond Binary: Gender Reassignment in a Case of 11β-Hydroxylase Deficiency.

authors

- Hithayathulla MA
- Putta Nagarajan HD
- Gopalakrishnan V
- Puliyadi Rishi K
- Chandrasekaran G

journal: Cureus

journalAbbr: Cureus

year: 2023

pubdate: 2023 Nov

volume: 15

issue: 11

pages: e48644

abstract: Congenital adrenal hyperplasia (CAH) encompasses a spectrum of disorders characterized by enzyme deficiencies in the hormone biosynthesis pathways of the adrenal glands, resulting in impaired cortisol synthesis. These disorders are typically inherited in an autosomal recessive pattern. Numerous enzymes participate in the hormonal synthesis within the adrenal glands, and the clinical presentation of affected individuals exhibits significant variability, contingent upon the specific enzyme deficiency and its severity. In this case, we present a compelling instance of 11β-hydroxylase deficiency (11βOHD). The patient initially presented as a male, with complaints of early-onset hypertension and intermittent hematuria. He had a history of precocious puberty and had experienced a progressive increase in breast size. Subsequently, the patient was found to have an XX karyotype, and a pelvic ultrasound revealed the presence of a uterus, two ovaries, and a rudimentary vagina. Gender reassignment surgery was done to this patient. This intricate case underscores the critical importance of promptly recognizing and effectively managing CAH. Timely and appropriate treatment is pivotal in ensuring the well-being of affected individuals.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10711344/

Record 469

pmcid: PMC10742254

pmid: 38136107

doi: 10.3390/children10121905

title: Beyond the Metabolic Syndrome: Non-Obvious Complications of Obesity in Children.

authors

- Kozioł-Kozakowska A
- Januś D
- Stępniewska A
- Szczudlik E
- Stochel-Gaudyn A
- Wójcik M

journal: Children (Basel, Switzerland)

journalAbbr: Children (Basel)

year: 2023

pubdate: 2023 Dec 9

volume: 10

issue: 12

pages:

abstract: Obesity is currently one of the most significant public health challenges worldwide due to the continuous increase in obesity rates among children, especially younger children. Complications related to obesity, including serious ones, are increasingly being diagnosed in younger children. A search was performed from January 2023 to September 2023 using the PubMed, Cochrane Library, Science Direct, MEDLINE, and EBSCO databases. The focus was on English-language meta-analyses, systematic reviews, randomized clinical trials, and observational studies worldwide. Four main topics were defined as follows: disorders of glucose metabolism; liver disease associated with childhood obesity; the relationship between respiratory disorders and obesity in children; and the effects of obesity on the hypothalamic-pituitary-gonadal axis and puberty. Understanding potential complications and their underlying mechanisms can expedite the diagnostic process and enhance the effectiveness of treatment. We aspire that this study will bring insight into the often-overlooked complications associated with obesity.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10742254/

Record 470

pmcid: PMC10215025

pmid: 37252506

doi: 10.7759/cureus.38149

title: Bilateral Gonadal Dysgerminoma in a Phenotypic Female With 46,XY Disorder of Sexual Development: A Case Report.

authors

- Pasquini Neto R
- Carnielli Tebet ML
- Ivanski Dória de Vasconcelos O
- Faucz Munhoz da Cunha M
- Figueroa Magalhães MC

journal: Cureus

journalAbbr: Cureus

year: 2023

pubdate: 2023 Apr

volume: 15

issue: 4

pages: e38149

abstract: The 46,XY disorder of sexual development (DSD) is a rare congenital condition characterized by a 46,XY karyotype associated with complete or disturbed female gonadal development and a non-virilized phenotype. The presence of Y chromosome material in these patients' karyotypes increases the risk of germ cell tumor development. The present study reports a unique case of a 16-year-old phenotypically female patient presenting with primary amenorrhea, who was later diagnosed with 46,XY DSD. After bilateral salpingo-oophorectomy, the patient was diagnosed with stage IIIC dysgerminoma. The patient received four cycles of

chemotherapy and showed a good response. The patient is currently alive and well, with no evidence of disease after the residual lymph node resection.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10215025/

Record 471

pmcid: PMC10044316

pmid: 36999117

doi: 10.1080/23320885.2023.2193273

title: Bilateral multicenter pseudohemangiomatous interstitial hyperplasia of the breast: a case report.

authors

- Yao C
- Liu C

journal: Case reports in plastic surgery & hand surgery

journalAbbr: Case Reports Plast Surg Hand Surg

year: 2023

pubdate: 2023

volume: 10

issue: 1

pages: 2193273

abstract: Bilateral multicenter breast pseudohemangiomatous stromal hyperplasia (PASH) is a rare, benign breast disease. Here, we report on a female patient with bilateral multicenter PASH who underwent a mastectomy and prosthetic reconstruction. The surgery was successful, and no recurrence was observed during the 18 months of follow up.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10044316/

Record 472

pmcid: PMC9853133

pmid: 36653041

doi: 10.1136/bcr-2022-251896

title: Bilateral primary breast Burkitt's lymphoma in pregnancy.

authors

- Altshuler E
- Wheeler S
- Daily K

journal: BMJ case reports

journalAbbr: BMJ Case Rep

year: 2023

pubdate: 2023 Jan 17

volume: 16

issue: 1

pages:

abstract: Pregnancy-associated cancer is defined as malignancy diagnosed during gestation or up to 1 year post partum. Treatment of cancer during pregnancy is complicated by the risk of harm to the fetus and limitations in safety data. Postpartum patients receiving chemotherapy, tyrosine-kinase inhibitors or hormonal agents should avoid breast feeding to avoid drug excretion in breast milk. Patients who will receive cytotoxic chemotherapy should be advised of the potential impact on their future fertility and offered fertility-preservation options. Breast cancer is the most common pregnancy-associated malignancy and is most frequently either invasive ductal or lobular carcinoma. Breast lymphoma is an exceedingly rare diagnosis that typically presents with unilateral disease in the seventh decade of life. Here, we present the case of a woman who presented with bilateral breast masses during the second trimester of pregnancy and was ultimately diagnosed with primary breast Burkitt's lymphoma.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9853133/

Record 473

pmcid: PMC10007813

pmid: 36906547

doi: 10.1186/s12884-023-05467-y

title: Body perception in pregnant women: a qualitative study.

authors

- Sohrabi Z
- Kazemi A
- Farajzadegan Z
- Janighorban M

journal: BMC pregnancy and childbirth

journalAbbr: BMC Pregnancy Childbirth

year: 2023

pubdate: 2023 Mar 11

volume: 23

issue: 1

pages: 165

abstract: BACKGROUND: Dramatic body changes in pregnancy cause severe concerns among pregnant women about their appearance. Therefore, this study aimed to explore body perception in pregnant women. MATERIALS AND METHODS: The qualitative study, using the conventional content analysis approach, was conducted on Iranian pregnant women who were in their second or third trimester of pregnancy. Participants were selected through purposeful sampling method. In-depth and semi-structured interviews were held with 18 pregnant women aged 22 to 36 years, using open-ended questions. Sampling was performed until data saturation was reached. RESULTS: Three main categories were extracted from 18 interviews: (1) "symbols," with two subcategories, including 'motherhood' and 'vulnerability,' (2) "feelings toward body changes," with five subcategories, including 'negative feelings toward skin changes,' 'feeling unfit,' 'attention-drawing body shape,' 'the ridiculous body shape' and 'obesity,' and (3) "attraction and beauty," with two subcategories, including 'sexual attraction' and 'facial beauty.' CONCLUSION: The results showed that pregnant women's body perception could be described as maternal feelings and feminine attitudes toward changes during pregnancy compared to mental ideals of facial and body beauty. It is recommended that Iranian women's body perception during pregnancy be evaluated using this study results and that counseling interventions be implemented for women with negative body perceptions.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10007813/

Record 474

pmcid: PMC10560982

pmid: 37708073

doi: 10.1148/rg.230014

title: Breast Imaging and Intervention during Pregnancy and Lactation.

authors

- Peterson MS
- Gegios AR
- Elezaby MA
- Salkowski LR
- Woods RW
- Narayan AK
- Strigel RM
- Roy M
- Fowler AM

journal: Radiographics : a review publication of the Radiological Society of North America, Inc

journalAbbr: Radiographics

year: 2023

pubdate: 2023 Oct

volume: 43

issue: 10

pages: e230014

abstract: Physiologic changes that occur in the breast during pregnancy and lactation create challenges for breast cancer screening and diagnosis. Despite these challenges, imaging evaluation should not be deferred, because delayed diagnosis of pregnancy-associated breast cancer contributes to poor outcomes. Both screening and diagnostic imaging can be safely performed using protocols based on age, breast cancer risk, and whether the patient is pregnant or lactating. US is the preferred initial imaging modality for the evaluation of clinical symptoms in pregnant women, followed by mammography if the US findings are suspicious for malignancy or do not show the cause of the clinical symptom. Breast MRI is not recommended during pregnancy because of the use of intravenous gadolinium-based contrast agents. Diagnostic imaging for lactating women is the same as that for nonpregnant nonlactating individuals, beginning with US for patients younger than 30 years old and mammography followed by US for patients aged 30 years and older. MRI can be performed for high-risk screening and local-regional staging in lactating women. The radiologist may encounter a wide variety of breast abnormalities, some specific to pregnancy and lactation, including normal physiologic changes, benign disorders, and malignant neoplasms. Although most masses encountered are benign, biopsy should be performed if the imaging characteristics are suspicious for cancer or if the finding does not resolve after a short period of clinical follow-up. Knowledge of the expected imaging appearance of physiologic changes and common benign conditions of pregnancy and lactation is critical for differentiating these findings from pregnancy-associated breast cancer. © RSNA, 2023 Online supplemental material is available for this article. Quiz questions for this article are available through the Online Learning Center.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10560982/

Record 475

pmcid: PMC10507758

pmid: 37732037

doi: 10.5114/aoms.2020.100637

title: Breast implant-associated anaplastic large cell lymphoma (BIA-ALCL) in Poland: analysis of patient series and practical guidelines for breast surgeons.

authors

- Pluta P
- Giza A
- Kolenda M
- Fendler W
- Braun M
- Chudobiński C
- Chałubińska-Fendler J
- Araszkiewicz M
- Loga K
- Lembas L
- Witkowska M
- Pluta A
- Kolasiński J
- Basta P
- Kołacińska-Voytkuv A
- Cieśla S
- Jesionek-Kupnicka D
- Jankau J
- Kuczyński M
- Kalinka E
- Zadrożny M
- Murawa D

journal: Archives of medical science : AMS

journalAbbr: Arch Med Sci

year: 2023

pubdate: 2023

volume: 19

issue: 5

pages: 1243-1251

abstract: INTRODUCTION: Although breast implant-associated anaplastic large cell lymphoma (BIA-ALCL) is infrequent, with less than 1000 noted cases worldwide, patients consenting for breast implant surgery should be aware of its risk. We describe the first Polish multicenter case-series data on BIA-ALCL patients and present diagnostic and treatment recommendation for breast surgeons. MATERIAL AND METHODS: In cooperation with the Polish Society of Surgical Oncology and Polish Lymphoma Research Group, we collected BIA-ALCL cases in Poland. RESULTS: We retrospectively reviewed clinical data of seven BIA-ALCL patients, diagnosed between July 2013 and November 2019. The median time from implant placement to the first BIA-ALCL symptoms was 65 months (range: 33-96 months). All the patients were exposed to textured implants at presentation. Capsulectomy with implant removal was performed in all the patients with immediate reimplantation in 2 cases. In a median follow-up of 19 months (range 5-81 months), there was no recurrence and all the patients stayed alive. Between 2013 and 2019, the incidence of BIA-ALCL in Polish female population age 30 and above ranged from 0 to 0.021/100 000/year. CONCLUSIONS: BIA-ALCL is scarce in the Polish population. In a short-term follow-up, patients' prognosis remains excellent. Due to the withdrawal of roughly textured implants from the market and the exclusion of likely the most potent etiologic factor, it might be expected that the incidence of BIA-ALCL will become even rarer.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10507758/

Record 476

pmcid: PMC10493074

pmid: 37700989

doi: 10.7759/cureus.43334

title: Breast Implant-Associated Anaplastic Large Cell Lymphoma Presenting in a Postpartum Patient: A Case Report.

authors

- Al-Shaikhli SA
- Mautner S
- Nasseri-Nik N
- Lampen-Sachar K

journal: Cureus

journalAbbr: Cureus

year: 2023

pubdate: 2023 Aug

volume: 15

issue: 8

pages: e43334

abstract: Breast implant-associated anaplastic large cell lymphoma (BIA-ALCL) is a rare cancer found in women with breast implants, occurring approximately eight years after their placement. Since the initial case report of BIA-ALCL in 1997, over 700 cases have been described worldwide and there is now a registry of cases diagnosed in the United States to help learn more about this disease process and prognosis. The majority of cases have been associated with Allergan's textured implants which have been recalled. As this disease is relatively rare with only 1,130 cases described worldwide, it is important to keep BIA-ALCL in mind in the differential diagnosis of a woman presenting with breast swelling and peri-implant fluid collection in the setting of having a textured implant. Outcomes for women with BIA-ALCL are favorable, as most disease is contained within the peri-implant capsule. Thus, complete surgical resection is often curative. Here, we report the case of a 40-year-old woman with BIA-ALCL who presented with unilateral breast swelling in the peripartum period.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10493074/

Record 477

pmcid: PMC10266515

pmid: 37325375

doi: 10.1097/GOX.0000000000005072

title: Breast Implant-associated Squamous Cell Carcinoma: Initial Review and Early Recommendations.

authors

- Glasberg SB
- Sommers CA
- McClure GT

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2023

pubdate: 2023 Jun

volume: 11

issue: 6

pages: e5072

abstract: UNLABELLED: The purpose of this study is to identify and describe all published cases of breast implant-associated squamous cell carcinoma (BIA-SCC) to gain a greater understanding of the incidence, presentation, diagnosis, treatment, and prognosis and to support development of recommendations that promote prompt diagnosis and management in clinical practice. METHODS: A scoping review of PubMed and social media sites was performed in August and September 2022 to identify published cases of SCC arising in the breast capsule. No limits were set on search results. Additional data review was begun on deidentified cases reported directly to American Society of Plastic Surgeons. RESULTS: Twelve articles met inclusion criteria and reported data on 16 total cases. Mean age of patients was 55.56 years (range, 40-81 years). Mean duration from initial implant placement to presentation was 23.56 years (range, 11-40 years). Cases occurred with silicone, saline, textured, and smooth implants. At the time of case publication or reporting, seven patients were alive, five were deceased and/or presumed deceased, and four were unreported. CONCLUSIONS: BIA-SCC seems to be a rare complication of breast implantation that can result in significant morbidity and mortality. Physicians should be aware of the presentation of BIA-SCC to promote prompt diagnosis and treatment. BIA-SCC should be discussed with all patients considering breast implantation as part of the informed-consent process.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10266515/

Record 478

pmcid: PMC10173349

pmid: 36357370

doi: 10.1177/10668969221102551

title: Breast Implanted-Associated Anaplastic Large Cell Lymphoma: A Case of Advanced Disease with Flow Cytometric Findings.

authors

- Demko N
- Safran T
- Vorstenbosch J
- Michel RP

journal: International journal of surgical pathology

journalAbbr: Int J Surg Pathol

year: 2023

pubdate: 2023 Jun

volume: 31

issue: 4

pages: 464-471

abstract: Breast implant-associated anaplastic large cell lymphoma (breast implant-associated ALCL) is a recently described, distinct clinicopathological entity associated with macrotextured breast implants. The diagnostic workup of a patient suspected to have breast implant-associated ALCL includes cytological assessment of effusions and tissue biopsies of any masses or enlarged lymph nodes, with morphologic and immunophenotypic evaluation and possible flow cytometric and molecular testing. We report the case of a woman found to have breast implant-associated ALCL on fine needle aspirate and core biopsy, who on surgical resection, had extensive local disease with involvement of the resection margins and lymph nodes, requiring

systemic treatment. We focus on the flow cytometric findings that identified a population of large cells on the CD30/side scatter dot plot and whose immunophenotype was consistent with breast implant-associated ALCL, highlighting the value of flow cytometry as an adjunct to morphological and immunophenotypic evaluation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10173349/

Record 479

pmcid: PMC9950848

pmid: 36843718

doi: 10.7759/cureus.34175

title: Breast Tuberculosis: A Case Report.

authors

- Pimentel Nunes M
- Branco Carvalho I
- Araújo I
- Almeida R
- Araújo J

journal: Cureus

journalAbbr: Cureus

year: 2023

pubdate: 2023 Jan

volume: 15

issue: 1

pages: e34175

abstract: Breast tuberculosis (BTB) is a rare manifestation of tuberculosis (TB), and it is more common in countries with a high incidence of TB. We describe a case of a 36-year-old Angolan woman, who had a history of breast reduction surgery, presenting with right breast enlargement, pain, purulent discharge through multiple skin openings, fever, and abdominal pain, progressively worsening in the past year. She had already undergone several surgical drainages and six months of treatment with ciprofloxacin, with no improvement. Breast ultrasound and MRI were performed, which revealed a large fluid collection, with several small abscesses and surrounding adenopathies, complicated by multiple fistulae. The fluid was drained through needle aspiration, which was found to be sterile for bacteria, mycobacteria, and fungi. A lymph node biopsy showed necrosis without granulomas, and the biopsy culture was positive for Mycobacterium tuberculosis (Mt). This case shows how a rare manifestation can simulate bacterial breast abscesses or cancer, and hence a high index of suspicion is necessary to reach the correct diagnosis and ensure appropriate treatment delivery in these patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9950848/

Record 480

pmcid: PMC10420177

pmid: 37569883

doi: 10.3390/ijms241512508

title: Can We Predict Prostate Cancer Metastasis Based on Biomarkers? Where Are We Now?

authors

- San Francisco IF

- Rojas PA
- Bravo JC
- Díaz J
- Ebel L
- Urrutia S
- Prieto B
- Cerda-Infante J

journal: International journal of molecular sciences

journalAbbr: Int J Mol Sci

year: 2023

pubdate: 2023 Aug 7

volume: 24

issue: 15

pages:

abstract: The incidence of prostate cancer (PC) has risen annually. PC mortality is explained by the metastatic disease (mPC). There is an intermediate scenario in which patients have non-mPC but have initiated a metastatic cascade through epithelial-mesenchymal transition. There is indeed a need for more and better tools to predict which patients will progress in the future to non-localized clinical disease or already have micrometastatic disease and, therefore, will clinically progress after primary treatment. Biomarkers for the prediction of mPC are still under development; there are few studies and not much evidence of their usefulness. This review is focused on tissue-based genomic biomarkers (TBGB) for the prediction of metastatic disease. We develop four main research questions that we attempt to answer according to the current evidence. Why is it important to predict metastatic disease? Which tests are available to predict metastatic disease? What impact should there be on clinical guidelines and clinical practice in predicting metastatic disease? What are the current prostate cancer treatments? The importance of predicting metastasis is fundamental given that, once metastasis is diagnosed, quality of life (QoL) and survival drop dramatically. There is still a need and space for more cost-effective TBGB tests that predict mPC disease.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10420177/

Record 481

pmcid: PMC10018749

pmid: 36936152

doi: 10.3389/fendo.2023.1101843

title: Case Report: Anastrozole as a monotherapy for pre-pubertal children with non-classic congenital adrenal hyperplasia.

authors

- Liu SC
- Suresh M
- Jaber M
- Mercado Munoz Y
- Sarafoglou K

journal: Frontiers in endocrinology

journalAbbr: Front Endocrinol (Lausanne)

year: 2023

pubdate: 2023

volume: 14

issue:

pages: 1101843

abstract: Most children with non-classic congenital adrenal hyperplasia (NC-CAH) due to 21-hydroxylase deficiency are asymptomatic and do not require cortisol replacement therapy unless they develop symptoms of hyperandrogenemia. The current practice is to treat symptomatic children with hydrocortisone aimed at suppressing excess adrenal androgen production irrespective of the child's level of endogenous cortisol production. Once on hydrocortisone therapy, even children with normal cortisol production require stress dosing. Some children with NC-CAH may present with premature adrenarche, growth acceleration, and advanced bone age, but with no signs of genital virilization and normal endogenous cortisol production. In these cases, an alternative therapy to hydrocortisone treatment that does not impact the hypothalamic-pituitary-adrenal axis, but targets increased estrogen production and its effects on bone maturation, could be considered. Aromatase inhibitors (AIs), which block the aromatization of androgen to estrogen, have been used off-label in men with short stature to delay bone maturation and as an adjunct therapy in children with classic CAH. The use of AI as a monotherapy for children with NC-CAH has never been reported. We present three pre-pubertal female children with a diagnosis of NC-CAH treated with anastrozole monotherapy after presenting with advanced bone age, early adrenarche, no signs of genital virilization, and normal peak cortisol in response to ACTH stimulation testing. Bone age z-scores normalized, and all three reached or exceeded their target heights. Monotherapy with anastrozole can be an effective alternative in slowing down bone maturation and improving height outcomes in children with NC-CAH and normal adrenal cortisol production.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10018749/

Record 482

pmcid: PMC10066061

pmid: 37012946

doi: 10.7759/cureus.35671

title: Central Precocious Puberty (CPP) in Two Girls With Autism Spectrum Disorder (ASD).

authors

- Moriuchi Y
- Fuchigami T
- Horie M
- Yamada R
- Morioka I

journal: Cureus

journalAbbr: Cureus

year: 2023

pubdate: 2023 Mar

volume: 15

issue: 3

pages: e35671

abstract: In recent years, some cases of central precocious puberty (CPP) have been reported in patients with autism spectrum disorder (ASD). Here, we report CPP in two girls with ASD. The first case was a girl, 7 years and 9 months of age. Breast budding was observed at 7 years and 2 months and pubic hair at 7 years and 8 months of age. She was diagnosed with CPP based on guidelines and ASD according to her developmental history. Considering the psychosocial burden caused by the discrepancy between her cognitive and behavioral

development and the progression of secondary sex characteristics, gonadotropin-releasing hormone (GnRH) analog therapy was initiated. Case 2 was a girl, 9 years and 8 months of age. She was diagnosed with ASD according to her developmental history. Treatment using oral aripiprazole for hypersensitivity to touch and taste had been initiated, with the onset of menarche at 9 years and 10 months of age. Breast budding had been observed before 7 years and 6 months old. She was diagnosed with CPP based on guidelines. Considering that menarche was not a significant psychosocial burden and the difficulty for her and her family to attend regular follow-ups, GnRH analog therapy was not initiated. Although the pathophysiological pathway linking ASD and CPP remains to be elucidated clinically, attention to CPP in ASD is needed, considering the increase in reported cases. In addition, the indication of GnRH analog therapy should be judged considering the psychosocial burden associated with secondary sexual characteristics.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10066061/

Record 483

pmcid: PMC10876265

pmid: 38380412

doi: 10.24911/SJP.106-1652031727

title: Central precocious puberty in Sudan: 10 years' experience of paediatric endocrinology centres.

authors

- Al-Hassan OA
- Limbe MS
- Warner J
- Musa SA
- Abdullah MA

journal: Sudanese journal of paediatrics

journalAbbr: Sudan J Paediatr

year: 2023

pubdate: 2023

volume: 23

issue: 2

pages: 145-152

abstract: Central precocious puberty (CPP) is frequently seen among cases presenting to our endocrine clinics. The purpose of this study was to have base line data on this condition with an attempt to point out any possible unique features of Sudan and to explore challenges faced in management and how that cultural and traditional practices may hamper care. Here, we are reporting the first data on this aspect from Sudanese subjects. Patients labelled as having CPP in Gafaar Ibnauf Children's Hospital and Soba University Hospital Endocrinology Clinics from January 2006 to 2016 are included in a descriptive hospital-based study which was conducted over 10 years in these two main paediatric endocrinology centres. Records of all patients with CPP were reviewed and challenges in diagnosis and management were identified. Most of the children with CPP presented late. Organic causes were more frequent among girls than what has been reported in the literature; in most boys, it was idiopathic. Almost half of the patients with underlying pathology were older than 6 years of age. Most cases including girls have an organic cause, thus magnetic resonance image should be done in all patients. Management of precocious puberty in a resource-limited country is faced with various challenges (e.g., expensive investigations and medications). We suggest establishing a unified protocol for managing these cases and creating collaboration between governmental, nongovernmental organisations and health services.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10876265/

Record 484

pmcid: PMC10642431

pmid: 37970614

doi: 10.4254/wjh.v15.i10.1109

title: Challenges and dilemmas in pediatric hepatic Wilson's disease.

authors

- Ghosh U
- Sen Sarma M
- Samanta A

journal: World journal of hepatology

journalAbbr: World J Hepatol

year: 2023

pubdate: 2023 Oct 27

volume: 15

issue: 10

pages: 1109-1126

abstract: Wilson disease is an autosomal recessive disorder affecting the ATP7B gene located on chromosome 13q. This leads to copper deposition in various organs, most importantly in the liver and brain. The genetic mutations are vast, well reported in the West but poorly documented in developing countries. Hence the diagnosis is made with a constellation of clinico-laboratory parameters which have significant overlap with other liver diseases and often pose a significant dilemma for clinicians. Diagnostic scoring systems are not fool-proof. The availability and affordability of chelators in developing countries impact the drug compliance of patients. While D-penicillamine is a potent drug, its side effects lead to drug discontinuation. Trientine is cost-prohibitive in developing countries. There is no single test to assess the adequacy of chelation. Exchangeable urinary copper is an essential upcoming diagnostic and prognostic tool. In the presence of cirrhosis, hypersplenism clouds the assessment of myelosuppression of drugs. Similarly, it may be difficult to distinguish disease tubulopathy from drug-induced glomerulonephritis. Neurological worsening due to chelators may appear similar to disease progression. Presentation as fulminant hepatic failure requires rapid workup. There is a limited window of opportunity to salvage these patients with the help of plasmapheresis and other liver-assisted devices. This review addresses the challenges and clinical dilemmas faced at beside in developing countries.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10642431/

Record 485

pmcid: PMC9633576

pmid: 36340228

doi: 10.1016/j.radcr.2022.10.002

title: Classic illustrations of benign and malignant phyllodes breast tumors in two patients.

authors

- Constante SA
- O'Connor S
- Jordan SG

journal: Radiology case reports

journalAbbr: Radiol Case Rep

year: 2023

pubdate: 2023 Jan

volume: 18

issue: 1

pages: 232-238

abstract: Phyllodes tumors, World Health Organization fibroepithelial tumors, are classified as benign, borderline, or malignant based on histopathology. Phyllodes must be distinguished from benign fibroadenomas, also WHO fibroepithelial tumors. The distinction of phyllodes from fibroadenomas can be challenging clinically, as these tumors may mirror one another. Here, we present 2 cases, classic clinical and imaging examples of benign and malignant phyllodes, to review the current epidemiology, classification, diagnosis, and treatment of phyllodes tumors.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9633576/

Record 486

pmcid: PMC9797444

pmid: 36194076

doi: 10.1097/PRS.0000000000009780

title: Clinical Implications and Management of Non-BIA-ALCL Breast Implant Capsular Pathology.

authors

- Vorstenbosch J
- Chu JJ
- Ariyan CE
- McCarthy CM
- Disa JJ
- Nelson JA

journal: Plastic and reconstructive surgery

journalAbbr: Plast Reconstr Surg

year: 2023

pubdate: 2023 Jan 1

volume: 151

issue: 1

pages: 20e-30e

abstract: The breast implant capsule is a dynamic structure that forms following the implantation of a device. Although normally benign, increased awareness of breast implant-associated anaplastic large-cell lymphoma (BIA-ALCL) highlights that disease may arise from the capsule. BIA-ALCL presents as a late seroma or mass but explains few of the late seromas found in breast implant patients. To date, many of these seromas lack a clear cause and are often described as "idiopathic." Several benign and malignant breast implant capsular diseases can cause a late seroma or mass, including breast implant-associated squamous cell carcinoma. Similar to early reports of BIA-ALCL, these conditions are rare and largely limited to case reports or series. The purpose of this special topic is to present a narrative review highlighting capsular abnormalities that contribute to the formation of late seroma or mass in an attempt to broaden the differential diagnosis and help plastic surgeons identify the cause. Specifically, we review the presentation and management of BIA-ALCL, synovial metaplasia, capsular epithelialization, late hematoma, double capsule, breast cancer, squamous cell carcinoma, mesenchymal tumor, and B-cell lymphoma. Although rare, plastic surgeons should consider these capsular conditions as causes of late seromas and masses. Usually, these conditions may be diagnosed by following the National Comprehensive Cancer Network screening guidelines for BIA-ALCL. Thorough evaluation and workup of late seromas and masses may lead to improved characterization of these rare breast implant capsular conditions and improve our understanding of their pathophysiology and management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9797444/

Record 487

pmcid: PMC10642698

pmid: 37917945

doi: 10.1097/AOG.0000000000005423

title: Combined Bilateral Salpingo-oophorectomy and Cesarean Delivery in BRCA1/2 Alteration Carriers : A Case Series.

authors

- Barker VE
- Vlachodimitropoulou E
- O'Brien P
- Iskaros J
- Rosenthal AN

journal: Obstetrics and gynecology

journalAbbr: Obstet Gynecol

year: 2023

pubdate: 2023 Dec 1

volume: 142

issue: 6

pages: 1500-1503

abstract: BACKGROUND: The cumulative lifetime risk of ovarian cancer is 16-68% and 11-30% in female BRCA1 and BRCA2 gene alteration carriers, respectively. Risk-reducing bilateral salpingo-oophorectomy (RRSO) is the only proven way to reduce ovarian cancer mortality. We report a series of patients who underwent risk-reducing surgery at the time of planned obstetric-indicated cesarean delivery. CASES: This is a case series of four women carrying a pathogenic germline BRCA1 or BRCA2 gene alteration who underwent RRSO at the time of cesarean delivery between March 1, 2018, and March 31, 2022. All women were referred during pregnancy to the University College London Hospitals Familial Cancer Clinic for consideration of RRSO at the time of obstetric-indicated cesarean delivery. Women were considered eligible for RRSO if they had a proven pathogenic germline alteration, would have completed childbearing after the cesarean delivery, and were older than age 35 or 40 years with BRCA1 or BRCA2 alterations, respectively. Operating time, blood loss, transfusion requirements, length of hospital stay, complications, and ability to breastfeed were assessed and, where possible, compared with the institutional means for similar patients who underwent cesarean delivery only, to determine whether RRSO was associated with increased morbidity. Women were contacted 11-59 months postprocedure to assess satisfaction. The mean blood loss was 687 mL (range 400-1,000 mL), mean operating time was 68 minutes, mean length of hospital stay was 3 days, and mean change in hemoglobin was -1 g/dL. No patient required a transfusion, had internal organ damage, returned to the operating room, or was readmitted. One of two women with intact breast tissue successfully breastfed, and the other chose to bottle feed. The mean contemporaneous institutional blood loss for cesarean delivery was not significantly different at 681 mL for singleton pregnancies and 872 mL for twin pregnancies. All four women reported a high level of satisfaction with the combined procedure. CONCLUSION: Our results show that RRSO can be performed at the time of cesarean delivery with high patient satisfaction. This approach can be offered to appropriately counseled individuals, with the benefit of avoiding the need for two separate procedures, with potentially reduced patient morbidity and health care costs.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10642698/

Record 488

pmcid: PMC10511348

pmid: 37746372

doi: 10.7759/cureus.43877

title: Comparative Efficacy of Mitchell's and Benson's Relaxation Techniques in Alleviating Pain and Improving Quality of Life Among Patients With Premenstrual Syndrome: A Randomized Controlled Trial.

authors

- Raipure A
- Patil S

journal: Cureus

journalAbbr: Cureus

year: 2023

pubdate: 2023 Aug

volume: 15

issue: 8

pages: e43877

abstract: Background Most women of reproductive age suffer physical discomfort or distress in the weeks before menstruation. Even though symptoms are typically not severe enough to impede daily activities seriously, they occasionally can. Physical problems like breast discomfort and bloating can also be a problem. The most irritating symptoms are those that affect mood and behaviour. Women of reproductive age experience premenstrual syndrome frequently, necessitating study into non-pharmacological methods for symptom reduction. Methodology The objective of the study was to evaluate and compare the clinical efficacy of Benson's relaxation technique to Laura Mitchell's physiological approach in patients with premenstrual syndrome. Study design was comparative parallel experimental study with patient-reported questionnaire data (online) collected before and post-intervention in tertiary hospital setting. Participants were 70 adult females aged 18 to 35 with premenstrual syndrome. Patients were administered Benson's relaxation technique once a day for a month versus Mitchell's relaxation technique once a day for a month. Both techniques were first taught for one session followed by home program to be performed by patients. The premenstrual syndrome questionnaire and a numerical pain rating scale were used to quantify premenstrual symptoms pre and post-intervention. Results The result revealed significant ($p<0.01$) improvement in premenstrual symptoms in both groups following the intervention. However, Bensons' relaxation technique was found to be more significant while alleviating the premenstrual symptoms. Conclusions When it comes to lowering the intensity of premenstrual syndrome in young people, Benson's relaxation method is superior to Mitchell's. Both approaches should be entrenched as a regular practice and can be employed on patients to improve their menstrual well-being.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10511348/

Record 489

pmcid: PMC10339479

pmid: 37442965

doi: 10.1186/s12887-023-04134-7

title: Cross-sectional study of characteristics of body composition of 24,845 children and adolescents aged 3-17 years in Suzhou.

authors

- Zhao Y
- Gong JX
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- Zhao XY

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journal: BMC pediatrics

journalAbbr: BMC Pediatr

year: 2023

pubdate: 2023 Jul 13

volume: 23

issue: 1

pages: 358

abstract: BACKGROUND: We aimed to analyze the characteristics of the body composition of children and adolescents aged 3-17 in Suzhou, China. METHODS: A cross-sectional study between January 2020 and June 2022 using bioelectrical impedance was conducted to determine the fat mass (FM), fat-free mass (FFM), skeletal muscle mass, and protein and mineral contents of 24,845 children aged 3-17 who attended the Department of Child and Adolescent Healthcare, Children's Hospital of Soochow University, China. Measurement data was presented in tables as mean ± SD, and groups were compared using the independent samples t-test. RESULTS: FM and fat-free mass increased with age in both boys and girls. The fat-free mass of girls aged 14-15 decreased after reaching a peak, and that of boys in the same age group was higher than that of the girls ($p < 0.05$). There were no significant differences in FM between boys and girls younger than 9- and 10-years old. The percentage body fat (PBF) and FM index of girls increased rapidly between 11 and 15 years of age ($p < 0.05$), and those of boys aged 11-14 were significantly lower ($p < 0.05$), suggesting that the increase in body mass index (BMI) was mainly contributed by muscle mass (MM) in boys. CONCLUSIONS: The body composition of children and adolescents varies according to their age and sex. A misdiagnosis of obesity made on the basis of BMI alone can be avoided if BMI is used in combination with FM index, percentage body fat, and other indexes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10339479/

Record 490

pmcid: PMC10075188

pmid: 37034116

doi: 10.1590/1984-3143-AR2023-0009

title: Cryopreservation of jaguar (Panthera onca) sperm cells using different cryoprotectants and different thawing temperatures.

authors

- Jorge-Neto PN
- Luczinski TC
- de Araújo GR
- Requena LA
- de Jesus RS
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- Zanella R
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journal: Animal reproduction

journalAbbr: Anim Reprod

year: 2023

pubdate: 2023

volume: 20

issue: 1

pages: e20230009

abstract: The cryopreservation of jaguar semen must be improved to produce high-quality biobanking doses. Until now, the rare studies of semen freezing in the species have only evaluated glycerol, always with a significant reduction in sperm quality in thawed semen. The purpose of this study was to assess the efficacy of three cryoprotectants, dimethylsulfoxide (DMSO), glycerol (GLY), and methanol (MET), in the cryopreservation of jaguar semen in an LDL-based extender, as well as the effect of thawing temperature on dosage quality. Five mature males with a history of reproduction were used. On the males, an infrared thermal image (IRT) was captured, the spicules and testes were analyzed, and the CASA system was used to evaluate the quality of fresh and thawed sperm. The superficial IRT was 4.6 ± 1.2 °C cooler than the anal sphincter, and the semen measured between 27.3 and 28.7 °C shortly after exiting the urethra. The total motility of fresh sperm was 55.3 ± 22.6%, and progressive motility was 36.3 ± 18%. The total motility of thawed sperm was 5.28 ± 2.51%, 4.49 ± %2.49, and 0.51 ± 0.62% for DMSO, GLY, and MET, respectively. DMSO and GLY performed better than MET, and there was no difference in thawing temperature (37°C 30 s vs. 50°C 12 s). All animals exhibit a considerable level of morphological changes in sperm. Low amounts of total and progressive motility were found in the thawed sperm. Males with a high level of sperm morphological changes were found to be fertile, but the lone male with normospermia was infertile. Thus, we contest the applicability of the commonly used morphological classification for bovines to felid species.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10075188/

Record 491

pmcid: PMC10455431

pmid: 37629572

doi: 10.3390/life13081715

title: D-Penicillamine-Induced Myasthenia Gravis-A Probable Complication of Wilson's Disease Treatment-A Case Report and Systematic Review of the Literature.

authors

- Antos A
- Członkowska A
- Bembenek J
- Kurkowska-Jastrzębska I
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journal: Life (Basel, Switzerland)

journalAbbr: Life (Basel)

year: 2023

pubdate: 2023 Aug 10

volume: 13

issue: 8

pages:

abstract: Wilson's disease (WD) is a genetic disorder with copper accumulation in various tissues leading to related clinical symptoms (mainly hepatic and neuropsychiatric) which can be in 85% of patients successfully

treated with anti-copper agents. However, during WD treatment neurological deterioration may occur in several patients. D-penicillamine (DPA) is one of the most frequently used drugs in WD treatment. Despite its efficacy, DPA can produce many adverse drug reactions, which should be recognized early. We present the case of a 51-year-old man diagnosed with the hepatic form of WD and initially treated with DPA in whom after 15 months of treatment, diplopia and evening ptosis occurred. WD treatment non-compliance as well as overtreatment were excluded. Supported by neurological symptoms, a positive edrophonium test, and high serum levels of antibodies against acetylcholine receptors (AChR-Abs), as well as low concentrations of antibodies against muscle-specific kinase (MuSK-Abs), the diagnosis of myasthenia gravis (MG), induced by DPA, was established. DPA was stopped; zinc sulfate for WD and pyridostigmine for MG symptoms were introduced. Diplopia and ptosis subsided after a few days, which supported our diagnosis. During a follow-up visit after 6 months, the patient did not present any MG symptoms. AChR-Abs level gradually decreased and MuSK-Abs were no longer detected. Pyridostigmine was stopped, and within 9 months of follow-up, the neurological symptoms of MG did not reoccur. The authors discussed the patient's neurological deterioration, performed a systematic review of DPA-induced MG in WD and concluded that MG is a rare and usually reversible complication of DPA treatment. DPA-induced MG generally occurs 2-12 months after treatment initiation and ocular symptoms predominate. Response to pyridostigmine treatment is good and MG symptoms usually reverse within one year after DPA treatment cessation. However, symptoms may persist in some cases where DPA treatment is only a trigger factor for MG occurrence.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10455431/

Record 492

pmcid: PMC10687328

pmid: 38034089

doi: 10.1093/sexmed/qfad054

title: Delayed, dramatic breast swelling in a transgender woman: a case report.

authors

- Szymanski K
- Munabi N
- Garcia M
- Ray E

journal: Sexual medicine

journalAbbr: Sex Med

year: 2023

pubdate: 2023 Oct

volume: 11

issue: 5

pages: qfad054

abstract: BACKGROUND: As the number of gender-affirming procedures performed in the United States increases, physicians caring for gender-nonconforming patients, regardless of practice location and focus, will likely encounter transgender women with breast implants. Increasingly, transgender women are seeking breast feminization. However, this population is less consistently receiving surveillance and routine breast care than cisgender women. AIM: This report aims to add to the growing body of knowledge addressing breast augmentation complications in transgender women and to highlight disparities in healthcare. METHODS: A case of breast implant-associated seroma at our institution was analyzed through chart review. A literature review was conducted using PubMed to gather all articles discussing breast implant-associated fluid collections in transgender patients. Prior to publication of this report, a Waiver of Consent was granted by the E.R.'s Institutional Review Board for the study under which this article was prepared. RESULTS: Our patient was an African American transgender woman presenting initially at age 60 with significant asymmetry due to dramatic

swelling of her left breast. The patient underwent bilateral breast augmentation outside of the United States 2 decades prior. The patient noted a gradual painless increase in her left breast size starting 3 years prior. She admitted that she was hesitant to seek a second opinion after being treated dismissively by another surgeon. Subsequent management included mammography and mirrored recommendations for late breast implant-associated seromas in cisgender patients: ultrasound, aspiration for cytology and culture, and removal of the implant and capsule. OUTCOMES: The fluid collection in our patient was determined to be a chronic hematoma and was managed surgically. Though this patient eventually achieved a good outcome, treatment was delayed due to barriers she faced as a transgender woman. CLINICAL IMPLICATIONS: Literature demonstrates that recommended management of late-onset breast-implant-associated seroma does not differ based on gender identity; however, transgender and GNC adults are more likely to receive less thorough care than cisgender women. Any patients undergoing breast augmentation with implants should be routinely evaluated for late complications, including seromas, which require prompt attention and methodical evaluation due to their potentially malignant nature. STRENGTHS AND LIMITATIONS: This article is limited in that it is a single report of breast seroma. It is strengthened by a PubMed review gathering all articles discussing breast-implant-associated fluid collections in transgender patients. CONCLUSION: We propose better education of physicians on how to care for transgender and gender-diverse patients should help mitigate the neglect and late presentation of such medical conditions in this vulnerable and marginalized population.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10687328/

Record 493

pmcid: PMC9863334

pmid: 36675445

doi: 10.3390/jcm12020517

title: Different Faces of Minipuberty in Preterm Twin Girls: A Case Report and Review of the Literature.

authors

- Pepe G
- Calafiore M
- Velletri MR
- Corica D
- Li Pomi A
- Wasniewska M
- Aversa T

journal: Journal of clinical medicine

journalAbbr: J Clin Med

year: 2023

pubdate: 2023 Jan 8

volume: 12

issue: 2

pages:

abstract: Minipuberty (MP) consists of a postnatal activation of the hypothalamic-pituitary-gonadal (HPG) axis, which occurs physiologically during the first months of life. In preterm infants, MP might lead to stronger hormonal stimulation, but specific literature is still scarce. We present the case of a pair of monochorionic diamniotic twin girls, born at 31 weeks of gestation and adequate for gestational age (AGA). At one month old, one of the twins presented with severe edema in the vulva and swelling of the major and minor labia. Laboratory evaluations highlighted increased LH, FSH and estradiol serum concentration. Pelvic ultrasonography and MRI showed a pubertal pattern. Brain imaging was unremarkable. During the one-year follow-up, a decreasing trend of hormonal levels was detected, together with the spontaneous regression of clinical and sonographic pubertal

signs. The same hormonal workup was also performed on the other twin, who displayed mildly elevated gonadotropins and estradiol, without evidence of pubertal clinical signs. This case suggests that the amplitude of postnatal HPG activation might be exacerbated in preterm infants, with evidence of puberty changes in clinical, laboratory and sonography data. The spontaneous resolution, together with the exclusion of other causes of precocious puberty, is suggestive for MP of infancy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9863334/

Record 494

pmcid: PMC9732114

pmid: 36505035

doi: 10.1016/j.jdcr.2022.11.010

title: Diffuse dermal angiomatosis associated with gestational gigantomastia.

authors

- Young KZ
- Briones NF
- Pedersen EA
- Fullen D
- Wang F

journal: JAAD case reports

journalAbbr: JAAD Case Rep

year: 2023

pubdate: 2023 Jan

volume: 31

issue:

pages: 49-52

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9732114/

Record 495

pmcid: PMC10870986

pmid:

doi:

title: E-Poster.

authors

journal: Indian Journal of Endocrinology and Metabolism

journalAbbr: Indian J Endocrinol Metab

year: 2023

pubdate: 2023 Dec

volume: 27

issue: Suppl 1

pages: S64-S121

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10870986/

Record 496

pmcid: PMC9870989

pmid: 36690835

doi: 10.1038/s41598-023-28602-3

title: Effect of gonadotropin-releasing hormone agonist monotherapy and combination therapy with growth hormone on final adult height in girls with central precocious puberty.

authors

- Cho AY
- Shim YS
- Lee HS
- Hwang JS

journal: Scientific reports

journalAbbr: Sci Rep

year: 2023

pubdate: 2023 Jan 23

volume: 13

issue: 1

pages: 1264

abstract: This study aimed to compare clinical parameters, including final adult height (FAH), in girls with central precocious puberty treated with gonadotropin-releasing hormone agonists (GnRHa) with and without growth hormone (GH). This retrospective study reviewed data of 210 girls with precocious puberty who had reached FAH in a long-term trial of GnRHa treatment. The subjects were divided into the GnRHa treatment group (n = 188), and the combined GnRHa + GH treatment group (n = 22). Chronological age, bone age, height, height standard deviation score, predicted adult height (PAH), FAH, Tanner stage, and hormone levels were assessed during the treatment period. At the start of treatment, PAH was 156.35 ± 6.34 cm in the GnRHa monotherapy group and 150.41 ± 5.32 cm in the GnRHa + GH group ($P < 0.001$). At the end of treatment, PAH was 166.25 ± 5.26 cm in the GnRHa group and 164.07 ± 4.99 cm in the combined GnRHa + GH treatment group, which had increased compared to the start of treatment. The FAH in the GnRHa group and GnRHa + GH combination group were 161.07 ± 4.78 cm and 159.63 ± 3.8 6 cm, respectively, without significant difference. In addition, the height gain (FAH-PAH) was significantly higher in the GnRHa + GH group than the GnRHa group (9.22 ± 6.03 cm vs. 4.72 ± 5.01 cm, $P < 0.001$). In girls with central precocious puberty, the height gain in the FAH compared to PAH at the start of treatment was significantly higher with the GnRHa + GH combination treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9870989/

Record 497

pmcid: PMC10249528

pmid: 37304979

doi: 10.4103/iju.iju_387_22

title: Effect of preoperative estrogen on complications after proximal hypospadias repair: A randomized controlled trial.

authors

- Pati AB

- Mishra P
- Mahalik SK
- Tripathy BB
- Mohanty MK

journal: Indian journal of urology : IJU : journal of the Urological Society of India

journalAbbr: Indian J Urol

year: 2023

pubdate: 2023 Apr-Jun

volume: 39

issue: 2

pages: 126-132

abstract: INTRODUCTION: Proximal hypospadias repair has many postoperative complications such as urethrocutaneous fistulae, wound dehiscence, and urethral stricture. The beneficial effect of estrogen to promote wound healing has been known. We designed a study to determine whether preoperative stimulation of tissue with estrogen can reduce the postoperative complications associated with wound healing in patients undergoing hypospadias repair. METHODS: Patients with proximal hypospadias requiring two-stage repairs (chordee correction followed by urethral tubularization) were randomized to estrogen and control groups before the second stage of surgery. In the former, topical estrogen cream (0.5 mg of estriol) was applied to the ventral penis for a month, whereas normal saline gel was applied to the latter; the urethroplasty was carried out thereafter. Patients were followed up for complications. RESULTS: There were 29 patients in the estrogen group and 31 in the placebo group after the exclusion criteria were met. There was no significant difference in the overall postoperative complications between the estrogen group (44.8%) and the placebo group (51.6%). The occurrence of urethrocutaneous fistula (37.9% vs. 51.6%) and dehiscence (41.4% vs. 45.2%) was not significantly different between the estrogen and placebo groups. Neourethral stricture occurred in four patients in the estrogen group, while none of the patients in the placebo group developed stricture. CONCLUSIONS: The preoperative application of topical estrogen cream to the ventral penis failed to demonstrate any significant effect on wound healing and complications.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10249528/

Record 498

pmcid: PMC8143058

pmid: 34031735

doi: 10.1007/s00266-021-02352-x

title: Effect of Reduction Mammoplasty on Insulin and Lipid Metabolism in the Postoperative Third month: Compensatory Hip Enlargement.

authors

- Bas S
- Oz K
- Akkus A
- Sizmaz M
- Serin E
- Durgun M
- Karsidag S

journal: Aesthetic plastic surgery

journalAbbr: Aesthetic Plast Surg

year: 2023

pubdate: 2023 Apr

volume: 47

issue: 2

pages: 536-543

abstract: BACKGROUNDS: The positive effects of reduction mammoplasty on metabolic profile have been shown in a limited number of studies. This study objective to reveal the effects of reduction mammoplasty on metabolic profile and anthropometric measurements. SUBJECTS AND METHOD: The study was prospectively conducted on 42 patients who were operated between April 2019 and March 2020. Fasting plasma glucose, fasting plasma insulin, total cholesterol, triglyceride, high-density lipoprotein and low-density lipoprotein cholesterol, HgA1c, homeostasis model assessment scores, adiponectin, leptin, and resistin levels were evaluated. In addition, age, height, weight, body mass index; breast, chest, waist, hip circumference; waist-hip ratio, and bilateral breast resection tissue weights were recorded. Data and blood samples were collected one hour before the operation, 6 and 12 weeks after the operation. RESULT: The patients' mean age was 43.14±10.24, and their average height was 159.42±4.96 cm. The excised bilateral dermo fatty tissue weight was 1435.85±721.16 g. At the postoperative 40th day a decrease in leptin ($p = 0.001$), resistin ($p = 0.008$), glucose ($p = 0.021$) and insulin resistance values ($p=0.013$) stated. There was an increase in adiponectin ($p < 0.001$) and HDL ($p = 0.013$) levels at the postoperative 40th day. In the postoperative third month, these data returned to the previous levels that were measured before operations. However, an increase in hip circumference ($p = 0.034$) and a decrease in waist-hip ratio ($p < 0.001$) was detected in third month. Also, there was no difference in body mass index and weight compared to pre-operation. CONCLUSION: After reduction mammoplasty, compensatory fat growth in the hip area, an increase in the hip circumference, and a decrease in the waist-hip ratio were observed in the postoperative third month. LEVEL OF EVIDENCE: This journal requires that authors assign a level of evidence to each article. For a full description of these Evidence-Based Medicine ratings, please refer to the Table of Contents or the online Instructions to Authors www.springer.com/00266 .

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8143058/

Record 499

pmcid: PMC9979010

pmid: 36857058

doi: 10.1001/jamasurg.2022.7625

title: Effect of Total Breast Reconstruction With Autologous Fat Transfer Using an Expansion Device vs Implants on Quality of Life Among Patients With Breast Cancer: A Randomized Clinical Trial.

authors

- Piatkowski AA
- Wederfoort JLM
- Hommes JE
- Schop SSJ
- Krastev TK
- van Kuijk SMJ
- van der Hulst RRWJ

journal: JAMA surgery

journalAbbr: JAMA Surg

year: 2023

pubdate: 2023 May 1

volume: 158

issue: 5

pages: 456-464

abstract: IMPORTANCE: There is a need for a new, less invasive breast reconstruction option for patients who undergo mastectomy in their breast cancer treatment. OBJECTIVE: To investigate quality of life (QoL) among patients undergoing a new breast reconstruction technique, autologous fat transfer (AFT), compared with that among patients undergoing implant-based reconstruction (IBR). DESIGN, SETTING, AND PARTICIPANTS: The BREAST trial was a randomized clinical trial conducted between November 2, 2015, and October 31, 2021, performed in 7 hospitals across the Netherlands. Follow-up was 12 months. Referrals could be obtained from general practitioners and all departments from participating or nonparticipating hospitals. The patients with breast cancer who had undergone mastectomy and were seeking breast reconstruction were screened for eligibility (radiotherapy history and physique) by participating plastic surgeons. Patients receiving postmastectomy radiotherapy were excluded. INTERVENTIONS: Breast reconstruction with AFT plus expansion or 2-phased IBR. Randomization was done in a 1:1 ratio. MAIN OUTCOMES AND MEASURES: The statistical analysis was performed per protocol. The predefined primary outcome was QoL at 12 months after final surgery. This was measured by the BREAST-Q questionnaire, a validated breast reconstruction surgery questionnaire. Questions on the BREAST-Q questionnaire are scored from 0 to 100, with a higher score indicating greater satisfaction or better QoL (depending on the scale). Secondary outcomes were breast volume and the safety and efficacy of the techniques. RESULTS: A total of 193 female patients (mean [SD] age, 49.2 [10.6] years) 18 years or older who desired breast reconstruction were included, of whom 91 patients in the AFT group (mean [SD] age, 49.3 [10.3] years) and 80 in the IBR group (mean age, 49.1 [11.0] years) received the allocated intervention. In total, 64 women in the AFT group and 68 women in the IBR group completed follow-up. In the IBR group, 18 patients dropped out mainly due to their aversion to implant use while in the AFT group 6 patients ended their treatment prematurely because of the burden (that is, the treatment being too heavy or tiring). The BREAST-Q scores were higher in the AFT group in all 5 domains and significantly higher in 3: satisfaction with breasts (difference, 9.9; P = .002), physical well-being: chest (difference; 7.6; P = .007), and satisfaction with outcome (difference, 7.6; P = .04). Linear mixed-effects regression analysis showed that QoL change over time was dependent on the treatment group in favor of AFT. The mean (SD) breast volume achieved differed between the groups (AFT: 300.3 [111.4] mL; IBR: 384.1 [86.6] mL). No differences in oncological serious adverse events were found. CONCLUSIONS AND RELEVANCE: This randomized clinical trial found higher QoL and an increase in QoL scores over time in the AFT group compared with the IBR group. No evidence was found that AFT was unsafe. This is encouraging news since it provides a third, less invasive reconstruction option for patients with breast cancer. TRIAL REGISTRATION: ClinicalTrials.gov Identifier: NCT02339779.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9979010/

Record 500

pmcid: PMC10543374

pmid: 37192767

doi: 10.1136/bmj-2022-074349

title: Effectiveness of spironolactone for women with acne vulgaris (SAFA) in England and Wales: pragmatic, multicentre, phase 3, double blind, randomised controlled trial.

authors

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journal: BMJ (Clinical research ed.)

journalAbbr: BMJ

year: 2023

pubdate: 2023 May 16

volume: 381

issue:

pages: e074349

abstract: OBJECTIVE: To assess the effectiveness of oral spironolactone for acne vulgaris in adult women. DESIGN: Pragmatic, multicentre, phase 3, double blind, randomised controlled trial. SETTING: Primary and secondary healthcare, and advertising in the community and on social media in England and Wales. PARTICIPANTS: Women (≥18 years) with facial acne for at least six months, judged to warrant oral antibiotics. INTERVENTIONS: Participants were randomly assigned (1:1) to either 50 mg/day spironolactone or matched placebo until week six, increasing to 100 mg/day spironolactone or placebo until week 24. Participants could continue using topical treatment. MAIN OUTCOME MEASURES: Primary outcome was Acne-Specific Quality of Life (Acne-QoL) symptom subscale score at week 12 (range 0-30, where higher scores reflect improved QoL). Secondary outcomes were Acne-QoL at week 24, participant self-assessed improvement; investigator's global assessment (IGA) for treatment success; and adverse reactions. RESULTS: From 5 June 2019 to 31 August 2021, 1267 women were assessed for eligibility, 410 were randomly assigned to the intervention (n=201) or control group (n=209) and 342 were included in the primary analysis (n=176 in the intervention group and n=166 in the control group). Baseline mean age was 29.2 years (standard deviation 7.2), 28 (7%) of 389 were from ethnicities other than white, with 46% mild, 40% moderate, and 13% severe acne. Mean Acne-QoL symptom scores at baseline were 13.2 (standard deviation 4.9) and at week 12 were 19.2 (6.1) for spironolactone and 12.9 (4.5) and 17.8 (5.6) for placebo (difference favouring spironolactone 1.27 (95% confidence interval 0.07 to 2.46), adjusted for baseline variables). Scores at week 24 were 21.2 (5.9) for spironolactone and 17.4 (5.8) for placebo (difference 3.45 (95% confidence interval 2.16 to 4.75), adjusted). More participants in the spironolactone group reported acne improvement than in the placebo group: no significant difference was reported at week 12 (72% v 68%, odds ratio 1.16 (95% confidence interval 0.70 to 1.91)) but significant difference was noted at week 24 (82% v 63%, 2.72 (1.50 to 4.93)). Treatment success (IGA classified) at week 12 was 31 (19%) of 168 given spironolactone and nine (6%) of 160 given placebo (5.18 (2.18 to 12.28)). Adverse reactions were slightly more common in the spironolactone group with more headaches reported (20% v 12%; p=0.02). No serious adverse reactions were reported. CONCLUSIONS: Spironolactone improved outcomes compared with placebo, with greater differences at week 24 than week 12. Spironolactone is a useful alternative to oral antibiotics for women with acne. TRIAL REGISTRATION: ISRCTN12892056.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10543374/

Record 501

pmcid: PMC10680311

pmid: 38008749

doi: 10.1186/s12905-023-02775-5

title: Effects of Pilates on inter-recti distance, thickness of rectus abdominis, waist circumference and abdominal muscle endurance in primiparous women.

authors

- Lee N
- Bae YH
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- Lee WH

journal: BMC women's health

journalAbbr: BMC Womens Health

year: 2023

pubdate: 2023 Nov 27

volume: 23

issue: 1

pages: 626

abstract: BACKGROUND: Pilates is expected to have a positive effect on women with weakened abdominal muscles after childbirth. Pilates may have a beneficial effect on the structure and function of the abdominal muscles in pregnant women. Therefore, the objective of this study was to investigate the effects of Pilates on inter-recti distance, thickness of the rectus abdominis, waist circumference, and abdominal muscle endurance in primiparous women. METHODS: Thirty-five primiparous postpartum women were assigned to either the Pilates exercise group (n = 20) or the control group (n = 15). Pilates was undertaken by the exercise group for 50 min/day, 5 days/week, for 4 weeks. The control group maintained their daily activities without any intervention. The inter-recti distance was measured at three locations along the linea alba, and the thickness of the rectus abdominis was measured using ultrasound. Abdominal muscle endurance was measured using a repeated 1-min curl-up test. Waist circumference was also measured. RESULTS: The exercise group showed significant improvements from baseline in inter-recti distance, waist circumference, and abdominal muscle endurance ($p < 0.05$). The control group showed no significant improvement in these variables. Compared with the control group, the exercise group showed significantly improved performance in terms of inter-recti distance, waist circumference, and abdominal muscle endurance ($p < 0.05$). CONCLUSIONS: The results of this study demonstrate that Pilates was effective in reducing inter-recti distance and waist circumference and improving abdominal muscle endurance in primiparous postpartum women. Pilates is considered an effective exercise for improving muscle structure and function in primiparous postpartum women, helping in the recovery from, and preventing, diastasis rectus abdominis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10680311/

Record 502

pmcid: PMC10093613

pmid: 37046457

doi: 10.3390/diagnostics13071239

title: Erdheim-Chester Disease of the Breast: First Review and First Case of Isolated Severe Gynecomastia.

authors

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journal: Diagnostics (Basel, Switzerland)

journalAbbr: Diagnostics (Basel)

year: 2023

pubdate: 2023 Mar 25

volume: 13

issue: 7

pages:

abstract: (1) Introduction: Erdheim-Chester disease (ECD) is a life-threatening condition and often a diagnostic challenge. It has recently been classified as a hematopoietic tumour, and the cases of ECD reported in the literature has dramatically increased during the last 15 years. (2) Methods: We describe the case of a 57-year-old male patient with severe gynecomastia, with a detailed description of his diagnostic iter and consequent surgical operation. We provide the first systematic review of the literature of breast involvement in ECD, following PRISMA guidelines, including 13 studies and 16 patients. (3) Results: Our report resulted to be the first case of gynecomastia as a single clinical and imaging feature of ECD described in English literature. A total of 81.3% of patients included were female. Among them, 76.9% had unilateral and nodular presentation, while male patients presented bilateral heterogeneous breast enlargement. Globally, 87.5% expressed breast alterations as their first manifestations of ECD. Only 50% presented skeletal involvement. (4) Conclusion: The reported case represents a unique addition to the literature. We found two different patterns in ECD-related breast involvement between male and female patients, an unusual M/F ratio, and a lower rate of bone involvement. Breast involvement is frequently the first clinical feature; therefore, breast caregivers should be aware of this dangerous and most likely underestimated condition.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10093613/

Record 503

pmcid: PMC10869008

pmid: 38371536

doi: 10.4103/ijd.ijd_539_23

title: Evaluation of Clinical, Hormonal and Ultrasound Parameters of Male Reproductive System in Leprosy: A Cross-Sectional Study.

authors

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- Kanathur S
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journal: Indian journal of dermatology

journalAbbr: Indian J Dermatol

year: 2023

pubdate: 2023 Nov-Dec

volume: 68

issue: 6

pages: 723

abstract: BACKGROUND AND OBJECTIVES: Testicular involvement in leprosy is neglected as it is insidious and silent, although it can have an impact on quality of life. Our study aimed at evaluating and estimating the frequency of the hormonal, radiological and clinical parameters in these patients. MATERIALS AND METHODS: A cross-sectional study was conducted including 31 male leprosy patients attending our OPD. After detailed clinical assessment, patients were subjected to ultrasonological with Doppler examination of the scrotum and hormonal evaluation by ELISA technique for serum follicle-stimulating hormone (FSH), luteinising hormone (LH) and total testosterone. RESULTS: Thirty-one patients were included with ages ranging from 19 to 54 with a mean of 34 years. Decreased libido (19.3%) was significantly observed in the abnormal FSH ($P < 0.01$) and testosterone ($P < 0.01$) groups. The majority of the clinical findings such as erectile dysfunction, infertility and altered pubic hair pattern were seen in the lepromatous spectrum. Six (19.6%) patients had increased LH and FSH. Twenty-three (74%) had testicular atrophy (<12 ml) as assessed by USG. There was a significant association between the bilateral decreased testicular volumes (TVs) and FSH ($P = 0.016$ on the left and $P < 0.01$ on the right). Four of 31 patients had altered testicular echo texture of which two belong to the BL spectrum. The increase in the resistive index (RI) corresponded significantly to the decreased TV ($P < 0.01$) albeit on the left side only. CONCLUSION: A sizeable proportion (74%) of the study sample had testicular atrophy. The significant association between increased RI and testicular atrophy, especially in patients with infertility, emphasises the ancillary role of Doppler indices in methodical diagnosis. Risk factors noted in the study include increased bacillary index, delayed initiation of treatment and recurrent ENL. Early detection, early initiation of MDT and specific therapies such as testosterone replacement can help improve the quality of life.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10869008/

Record 504

pmcid: PMC9851055

pmid: 36686607

doi: 10.2147/CCID.S368273

title: Exploration of Influencing Factors for Postoperative Recurrence in Patients with Madelung's Disease on the Basis of Multivariate Stepwise Cox Regression Analysis.

authors

- Li S
- Xiao Y
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journal: Clinical, cosmetic and investigational dermatology

journalAbbr: Clin Cosmet Investig Dermatol

year: 2023

pubdate: 2023

volume: 16

issue:

pages: 103-110

abstract: PURPOSE: Madelung's disease (MD) is a rare condition of massive deposits of fat accumulations between superficial and deep fascia at typical locations. There is an absence of systematic studies related to MD in the Chinese cohort. Thus, the objective of the study was to investigate the clinical features of the MD cases in our institution and to explore the clinical variables associated with postoperative recurrence. MATERIALS AND METHODS: We retrospectively analyzed the clinical information of 21 individuals with MD

from 2013 to 2021 enrolled in our institution. The paired t -test and χ 2 test were, respectively, used to determine the difference between continuous and classified variables. The univariate Kaplan-Meier analysis by log-rank and multivariate stepwise Cox regression analysis were used to explore variables possibly associated with postoperative recurrence in MD individuals. RESULTS: In the current study, 90.48% of the studied patients were male with a mean age of 48.76 years old. About 61.90% exhibited type I MD. MD patients who experienced postoperative recurrence had a higher age, BMI, incidence of chronic complications, and prevalence of alcoholism than the other MD patients without recurrence (P < 0.05). The univariate Kaplan-Meier analysis by log-rank identified that age, BMI, alcoholism, and comorbidities were influencing factors related with postoperative recurrence (P < 0.05). CONCLUSION: Demographic characteristics of the 21 studied Chinese cases with MD were generally in accordance with previously published data of other foreign populations. The factors possibly influencing the postoperative recurrence for patients with MD were age, BMI, alcoholism, and a combination of comorbidities. This is the first time that a summarization of clinical characteristics and postoperative recurrence variables of Chinese patients with MD has been reported.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9851055/

Record 505

pmcid: PMC10576758

pmid: 37838805

doi: 10.1038/s41598-023-44808-x

title: Exploring enablers and barriers to breast self-examination among women in the North Shewa Zone, Oromia, Ethiopia: a qualitative study.

authors

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- Gashaw K
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journal: Scientific reports

journalAbbr: Sci Rep

year: 2023

pubdate: 2023 Oct 14

volume: 13

issue: 1

pages: 17455

abstract: Breast cancer (BC) is the leading cause of cancer death worldwide and the second most common cancer overall. Breast self-examination (BSE) is one of the cheapest methods used for the early detection of BC in asymptomatic women. More than 90% of cases of BC can be detected by women themselves. This reality stresses the importance of BSE as the key BC detection mechanism. However, in Ethiopia, most of the BE studies were not conducted among women in the general population. Therefore, this study aimed to explore enablers and barriers to breast self-examination among women in the North Shewa Zone, Oromia. A descriptive qualitative study design was conducted from August 1, 2022, to September 30, 2022. Five focus group discussions (FGDs) were conducted with 46 women from one selected district in the North Shewa Zone, Oromia. A Purposive sampling technique was used to select participants for FGD. The audio-recorded data were transcribed verbatim to "Afan Oromo". Transcribed data were translated into English. The data were manually coded into themes and analyzed manually by using inductive thematic analysis. The findings of the study were discussed under five themes of enablers and three themes of barriers. The five themes of enablers were knowledge about BC, knowledge about BSE, experience of BSE practice, perceived susceptibility, and

perceived benefit of BSE practice. The four themes of barriers were low knowledge of BSE practice, misconceptions about BSE practice, and fear of detecting BC. These findings suggest that targeted health education programs, collaboration between healthcare providers and local stakeholders, and the availability of support services can play a crucial role in overcoming barriers and encouraging BSE practice for early detection of breast abnormalities.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10576758/

Record 506

pmcid: PMC10744413

pmid: 38138197

doi: 10.3390/medicina59122094

title: Exploring Hormone Therapy Effects on Reproduction and Health in Transgender Individuals.

authors

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journal: Medicina (Kaunas, Lithuania)

journalAbbr: Medicina (Kaunas)

year: 2023

pubdate: 2023 Nov 29

volume: 59

issue: 12

pages:

abstract: Transgender individuals often face elevated mental health challenges due to gender dysphoria, but gender-affirming treatments such as surgery and hormone therapy have been linked to significant improvements in mental well-being. The potential influence of time and circadian rhythms on these treatments is prevalent. The intricate interplay between hormones, clock genes, and fertility is profound, acknowledging the complexity of reproductive health in transgender individuals. Furthermore, risks associated with gender-affirming hormonal therapy and potential complications of puberty suppression emphasize the importance of ongoing surveillance for these patients and the need of fertility preservation and family-building options for transgender individuals. This narrative review delves into the intricate landscape of hormone therapy for transgender individuals, shedding light on its impact on bone, cardiovascular, and overall health. It explores how hormone therapy affects bone maintenance and cardiovascular risk factors, outlining the complex interplay of testosterone and estrogen. It also underscores the necessity for further research, especially regarding the long-term effects of transgender hormones. This project emphasizes the critical role of healthcare providers, particularly obstetricians and gynecologists, in providing affirming care, calling for comprehensive understanding and integration of transgender treatments. This review will contribute to a better understanding of the impact of hormone therapy on reproductive health and overall well-being in transgender individuals. It will provide valuable insights for healthcare providers, policymakers, and transgender individuals themselves, informing decision-making regarding hormone therapy and fertility preservation options. Additionally, by identifying research gaps, this review will guide future studies to address the evolving healthcare needs of transgender individuals. This project represents a critical step toward addressing the complex healthcare needs of this population. By synthesizing existing knowledge and highlighting areas for further investigation, this review aims to improve the quality of care and support provided to transgender individuals, ultimately enhancing their reproductive health and overall well-being.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10744413/

Record 507

pmcid: PMC10278025

pmid: 37342480

doi: 10.1089/trgh.2021.0130

title: Extended Use of Histrelin Implant in Pediatric Patients.

authors

- Pine-Twaddell E
- Newfield RS
- Marinkovic M

journal: Transgender health

journalAbbr: Transgend Health

year: 2023

pubdate: 2023 Jun

volume: 8

issue: 3

pages: 264-272

abstract: PURPOSE: Histrelin implant (HI) is a gonadotropin-releasing hormone agonist (GnRHa) used in pediatrics to treat central precocious puberty (CPP) and for pubertal suppression in transgender/non-binary (TG/NB) youth with gender dysphoria. HI is designed for annual removal/replacement; however, effectiveness has been reported beyond 1 year. No previous study has assessed prolonged HI use in TG/NB youth. We hypothesize that HI is effective >12 months in TG/NB youth as described in children with CPP. METHODS: This retrospective, two-center study included 49 subjects with 50 HI retained ≥17 months, in TG/NB (42) and CPP (7). Pubertal suppression was assessed biochemically and/or clinically (testicular/breast exams). Escape from pubertal suppression and HI removal is also characterized. RESULTS: Most implants (42/50) maintained clinical/biochemical suppression for the duration of the study. The average use of a single HI was 37.5±13.6 months. Pubertal suppression escape occurred in eight subjects at average 30.4 months from placement: five had only biochemical; two clinical; and one both clinical and biochemical escape. After an average of 32.9 months, only 3/23 HI removed had adverse effects (HI broken, difficult removal). CONCLUSION: Extended use of HI in our TG/NB and CPP subjects was efficacious, resulting in sustained biochemical and clinical pubertal suppression in most. Suppression escape occurred at 15-65 months. Complications at HI removal were infrequent. Keeping HI for extended time would improve cost and morbidity, while maintaining efficacy and safety for most patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10278025/

Record 508

pmcid: PMC9843029

pmid: 36694785

doi: 10.12669/pjms.39.1.6647

title: Factors leading to delayed and challenging presentation of benign breast lumps in young females.

authors

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journal: Pakistan journal of medical sciences

journalAbbr: Pak J Med Sci

year: 2023

pubdate: 2023 Jan-Feb

volume: 39

issue: 1

pages: 80-85

abstract: BACKGROUND AND OBJECTIVE: A delayed presentation of benign breast diseases may lead to a therapeutic challenge when they enlarge enormously or become multiple. Aim of this study was to evaluate the factors leading to delayed presentation of benign breast lumps. METHODS: This cross-sectional study was conducted at Madinah Teaching Hospital and Allied Hospital, Faisalabad from January 2019 to October 2021. One hundred and forty five female patients were selected by non-probability purposive sampling. Patients with large size (>5cm) or multiple benign breast lumps were included. An interview was conducted using structured questionnaire translated in Urdu. Factors leading to delayed presentation and social impact scale for stigma were evaluated. RESULTS: Patients had a mean age of 26.52 ± 6.90 years. The average delay of seeking medical care was 8.48 ± 8.41 months. Factors leading to delayed presentation were; lack of knowledge n=112 (77.2%) and parda (religious issues) n=112 (77.2%), followed by poverty n=109 (75.2%), and fear of cancer n=90 (62.1%). All the sub-scales of stigma, i.e., social rejection, financial insecurity, internalized shame and social isolation were high in late presenters, though, only financial insecurity was significantly high in late presenters (p=0.03). CONCLUSION: Lack of awareness, socioeconomic factors and disease related stigma were the main factors related to delayed presentation in young females with benign breast diseases. Addressing these factors may improve timely diagnosis and management of delayed and challenging cases.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9843029/

Record 509

pmcid: PMC10758266

pmid: 38164309

doi: 10.7759/cureus.49792

title: Factors That Affect the Likelihood of Undergoing Cosmetic Procedures Among the General Population of Western Region, Saudi Arabia: A Cross-Sectional Study.

authors

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journal: Cureus

journalAbbr: Cureus

year: 2023

pubdate: 2023 Dec

volume: 15

issue: 12

pages: e49792

abstract: BACKGROUND AND AIM: Cosmetic procedures are surgical and non-surgical procedures that improve and reshape body or facial structures to improve someone's appearance, self-esteem, and confidence. In recent years, these procedures have gained more popularity, and both the number of procedures performed and the cosmetic procedure market are growing dramatically worldwide. The objective of our research is to carry out a cross-sectional investigation to assess the factors that affect the likelihood of undergoing cosmetic surgery in the Western Region of Saudi Arabia. METHODS: In this study, a descriptive, cross-sectional methodology was employed. The intended sample includes residents of Makkah, Medina, Jeddah, and Al-taif cities who are 18 years of age or older, representing the general population. Data collection was carried out through an online questionnaire created using Google Forms, which was disseminated electronically via social media platforms. The questionnaire gathered demographic information and questions that evaluate the time spent on social media, likelihood of having cosmetic surgery, cosmetic surgery experience, social media exposure, personal experience, and self-rating of attractiveness. RESULTS: The study included a total of 507 participants, with 389 (76.7%) being female and 118 (23.3%) being male. In our sample, there was a significant correlation between the likelihood of undergoing cosmetic surgery in women who have a longer exposure to media and lower self-rating of attractiveness. For men, media exposure and previous cosmetic procedures were significantly affecting their likelihood of undergoing cosmetic procedures. CONCLUSIONS: Females who had a lower self-rating of attractiveness and a longer exposure to social media were more likely to undergo a cosmetic procedure. However, to gain a more comprehensive understanding, further research should be conducted.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10758266/

Record 510

pmcid: PMC10555117

pmid:

doi: 10.1210/jendso/bvad114.246

title: FRI251 A Combination Of Klinefelter Syndrome And The Classic Form Of Congenital Dysfunction Of The Adrenal Cortex: Clinical Case.

authors

- Volkova NI

journal: Journal of the Endocrine Society

journalAbbr: J Endocr Soc

year: 2023

pubdate: 2023 Oct 5

volume: 7

issue: Suppl 1

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10555117/

Record 511

pmcid: PMC10553959

pmid:

doi: 10.1210/jendso/bvad114.1261

title: FRI326 Hypophysitis: A Formidable Diagnostic Challenge.

authors

- Pandey S
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journal: Journal of the Endocrine Society

journalAbbr: J Endocr Soc

year: 2023

pubdate: 2023 Oct 5

volume: 7

issue: Suppl 1

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10553959/

Record 512

pmcid: PMC10352531

pmid: 37469658

doi: 10.1155/2023/9279934

title: Gestational Pseudoangiomatous Stromal Hyperplasia Presenting as Gigantomastia: A Case Report of a Rare Breast Entity with Clinical Recommendations by a Multidisciplinary Team.

authors

- Wang SJ
- Maheswaran S
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journal: Case reports in surgery

journalAbbr: Case Rep Surg

year: 2023

pubdate: 2023

volume: 2023

issue:

pages: 9279934

abstract: INTRODUCTION: Pseudoangiomatous stromal hyperplasia (PASH) presenting as gigantomastia is rare in pregnancy but can result in severe clinical consequences for both mother and fetus. Case Presentation . A 43-year-old female with a history of biopsy-proven bilateral PASH presented at 22 3/7 weeks gestation with massive bilateral breast enlargement that was symptomatic. After multidisciplinary care, she underwent bilateral mastectomies and delivered at term with no additional complications. CONCLUSION: Pregnant women who undergo mastectomies for PASH-induced gigantomastia during their second trimesters will likely recover quickly, and fetal risks are low. Given the rarity of this breast entity, management guidelines are sparse. Our case report is an effort to comprehensively review this condition and share the clinical recommendations made

by our institution's multidisciplinary team.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10352531/

Record 513

pmcid: PMC10003210

pmid: 36902642

doi: 10.3390/jcm12051855

title: Giant Juvenile Fibroadenoma: Case Report and Review of the Literature.

authors

- Eleftheriades A
- Tsarna E
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journal: Journal of clinical medicine

journalAbbr: J Clin Med

year: 2023

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volume: 12

issue: 5

pages:

abstract: Fibroadenomas are common benign breast tumors. Fibroadenomas that exceed 5 cm in diameter, weigh more than 500 g, or replace more than four-fifths of the breast are characterized as giant. A fibroadenoma diagnosed in patients during childhood or adolescence is characterized as juvenile. An extensive PubMed search of the literature in English up until August 2022 was performed. In addition, a rare case of a gigantic fibroadenoma in an 11-year-old premenarchal girl who was referred to our adolescent gynecology center is presented here. Eighty-seven cases of giant juvenile fibroadenomas have been reported in the literature along with our case. Patients with giant juvenile fibroadenoma presented at a mean age of 13.92 years and usually after menarche. Juvenile fibroadenomas are usually unilateral, occurring either in the right or the left breast; the majority of them are diagnosed when they are already more than 10 cm in size, and they are most frequently treated with total lump excision. Differential diagnosis includes phyllodes tumors and pseudo-angiomatous stromal hyperplasia. Conservative management is feasible, but surgical excision is recommended to patients with suspicious imaging features or when the mass grows rapidly.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10003210/

Record 514

pmcid: PMC10210576

pmid: 37092627

doi: 10.23750/abm.v94i2.14028

title: Gynecomastia: an uncommon, destabilizing condition of the male adolescent. our therapeutic choice.

authors

- Pinelli M
- De Maria F
- Ceccarelli P
- Pedrieri B
- Bianchini MA
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- De Santis G

journal: Acta bio-medica : Atenei Parmensis

journalAbbr: Acta Biomed

year: 2023

pubdate: 2023 Apr 24

volume: 94

issue: 2

pages: e2023055

abstract: Gynecomastia is the benign enlargement of breast's the glandular tissue in male population. Gynecomastia can involve fatty and/or glandular tissue. At the basis of pediatric gynecomastia there is a multifactorial imbalance in the ratio of estrogen to androgens tissue levels. In more than 95% of the cases gynecomastia development is idiopathic. Secondary causes of gynecomastia in adolescents are relatively rare (less than 5%) and may arise from uncommon pathological conditions. Gynecomastia is self-limited and regresses in 1-3 years in 84%, 47% and 20% of adolescents with mild, moderate and severe gynecomastia. The correct first line of therapy is observation and reassurance in the treatment of mild cases. In order to manage adolescent gynecomastia is advised to adopt a tailored therapy. Despite gynecomastia is a common condition only few adolescents need cosmetic or antalgic treatment. Medical therapy should be considered in patient with emotional distress or psychological limitation on normal activities. Finally, if gynecomastia does not go in remission after two years surgical procedures should be performed. The aim of this article is to be an updated discussion of pubertal gynecomastia in every way and report our surgical experience with a retrospective study. In conclusion surgical treatment of this condition is a quiet rare procedure but, in according to global literature we demonstrated that it is a safe surgery with low rate of complications.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10210576/

Record 515

pmcid: PMC10464010

pmid: 37620840

doi: 10.1186/s12887-023-04209-5

title: Hepatic adenoma in a 7-year-old girl: a case report and literature review.

authors

- Gao Y
- Zhou J
- Xie YC
- Qiu LJ
- Duan L
- A ZX
- Wu HF
- Lv MX

journal: BMC pediatrics

journalAbbr: BMC Pediatr

year: 2023

pubdate: 2023 Aug 24

volume: 23

issue: 1

pages: 420

abstract: BACKGROUND: Hepatocellular adenomas (HCAs) are rare benign tumors of the liver that occur predominantly in women taking oral contraceptives. In children, HCAs comprise < 5% of hepatic tumors. We report a case of HCAs in a 7-year-old girl with estrogen and glucose imbalance. CASE PRESENTATION: A 7-year-old girl was presented to our hospital with bilateral breast enlargement for 2 months, polydipsia, polyuria, polyphagia, hyperglycemia, and significant weight gain. Computed tomography (CT) showed a 7.2 cm×6.9 cm×5.3 cm round-shaped mass in the left inner lobe of the liver, ovarian ultrasound showed multiple follicles in the ovaries bilaterally, and cranial magnetic resonance imaging (MRI) showed an enlarged superior pituitary. Hematological and biochemical results were as follows: fasting glucose was 19.7 mmol/L, estradiol was 122.9 pmol/L, follicle-stimulating hormone 10.81 IU/L, luteinizing hormone 10.99 IU/L, insulin-like growth factor 1,513 ng/mL, glutamine aminotransferase 86 U/L, and alkaline phosphatase 362 U/L. Thyroid functions, methemoglobin, fetal protein, carcinoembryonic antigen, and chorionic gonadotropin were normal. The patient had a complete surgical resection of the liver tumor, and the postoperative histopathological diagnosis was HCAs. After the surgery, insulin was injected and the glucose levels were stable. During the 36-month follow-up period, neither tumor recurrence nor significant abnormalities were detected using color Doppler ultrasound of the liver. The child's precocious puberty is currently under control. CONCLUSIONS: HCAs are particularly rare in children with liver tumors, and risk factors for the development of HCAs in children include sex hormone imbalance, obesity, Fanconi anemia (FA), glycogen storage diseases (GSDs) type I, III, and IV, galactosemia, immunodeficiency, congenital portosystemic shunts (CPSS), cardiac hepatopathy status-post Fontan procedure, Hurler syndrome, familial adenomatous polyposis, germline HNF1A mutations, and maturity-onset diabetes of the young type 3. Most HCAs are detected during a physical examination without clinical symptoms, and some patients may present with symptoms such as abdominal pain, abdominal distension, and abdominal masse. Serum liver function tests can show increased alkaline phosphatase (ALP) and γ- glutamyl transferase (GT), whereas α-Fetoprofein (AFP) levels are normal. The definitive diagnosis relies mainly on histopathological examination. Because HCAs can rupture and bleed and become malignant. Early surgical treatment is recommended after detection.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10464010/

Record 516

pmcid: PMC10728840

pmid: 38116543

doi: 10.5812/ijpr-130336

title: Hepatotoxicity Induced by Azole Antifungal Agents: A Review Study.

authors

- Rakhshan A
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journal: Iranian journal of pharmaceutical research : IJPR

journalAbbr: Iran J Pharm Res

year: 2023

pubdate: 2023 Jan-Dec

volume: 22

issue: 1

pages: e130336

abstract: CONTEXT: Fungal infections are very common, and several medications are used to treat them. Azoles are prescribed widely to treat fungal infections. In addition to therapeutic effects, any drug can be accompanied by side effects in patients. One of the most important complications in this regard is liver injury. Therefore, hepatotoxicity induced by azole antifungal drugs were reviewed in this study. EVIDENCE ACQUISITION: English scientific papers were evaluated to review the effects of hepatotoxicity by azole antifungal agents, and the related studies' results were summarized using a table. The systematic search was implemented on electronic databases, including PubMed, Google Scholar, and Science Direct. Original articles and review articles that were published before April 1, 2022, were included in the study. Those articles without available full text or non-English articles were excluded. Also, articles that reported pediatric data were excluded. RESULTS: Most studies have reported the effects of hepatotoxicity by azole antifungal agents, and their mechanisms have been described. CONCLUSIONS: Clinical evaluations regarding the hepatotoxicity of antifungal agents provided in the literature were reviewed. Therefore, it is recommended to prescribe these drugs with caution in high-risk patients suffering from liver diseases, and patients should be monitored for hepatotoxicity. However, more research is needed to evaluate the hepatotoxicity of azole antifungal agents and select appropriate drugs according to cost-effectiveness and the side effects' profiles, relying on lower incidence of this liver complication.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10728840/

Record 517

pmcid: PMC9955103

pmid: 36832611

doi: 10.3390/e25020245

title: Homogeneous Adaboost Ensemble Machine Learning Algorithms with Reduced Entropy on Balanced Data.

authors

- Ramakrishna MT
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- Izonin I
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journal: Entropy (Basel, Switzerland)

journalAbbr: Entropy (Basel)

year: 2023

pubdate: 2023 Jan 29

volume: 25

issue: 2

pages:

abstract: Today's world faces a serious public health problem with cancer. One type of cancer that begins in the breast and spreads to other body areas is breast cancer (BC). Breast cancer is one of the most prevalent cancers that claim the lives of women. It is also becoming clearer that most cases of breast cancer are already advanced when they are brought to the doctor's attention by the patient. The patient may have the evident lesion removed, but the seeds have reached an advanced stage of development or the body's ability to resist them has weakened considerably, rendering them ineffective. Although it is still much more common in more developed nations, it is also quickly spreading to less developed countries. The motivation behind this study is to use an ensemble method for the prediction of BC, as an ensemble model aims to automatically manage the

strengths and weaknesses of each of its separate models, resulting in the best decision being made overall. The main objective of this paper is to predict and classify breast cancer using Adaboost ensemble techniques. The weighted entropy is computed for the target column. Taking each attribute's weights results in the weighted entropy. Each class's likelihood is represented by the weights. The amount of information gained increases with a decrease in entropy. Both individual and homogeneous ensemble classifiers, created by mixing Adaboost with different single classifiers, have been used in this work. In order to deal with the class imbalance issue as well as noise, the synthetic minority over-sampling technique (SMOTE) was used as part of the data mining pre-processing. The suggested approach uses a decision tree (DT) and naive Bayes (NB), with Adaboost ensemble techniques. The experimental findings shown 97.95% accuracy for prediction using the Adaboost-random forest classifier.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9955103/

Record 518

pmcid: PMC10040029

pmid: 36963764

doi: 10.1136/bcr-2022-254429

title: Hypothalamic hamartoma: a cause of precocious puberty.

authors

- Castro C
- Machado Morais J
- Correia AL
- Espada F

journal: BMJ case reports

journalAbbr: BMJ Case Rep

year: 2023

pubdate: 2023 Mar 24

volume: 16

issue: 3

pages:

abstract: Hypothalamic hamartomas are uncommon congenital malformations that present as precocious puberty, gelastic seizures and/or psychiatric disorders. Characteristic changes in MRI scans lead to a diagnosis. Treatment may include surgery or gonadotropin-releasing hormone agonists (GnRHa) depending on clinical manifestations.Here, we describe a case of hypothalamic hamartoma diagnosed in a girl in middle childhood, who presented with early development of secondary sexual characteristics. Physical examination, hormonal study, bone age and pelvic ultrasound findings were consistent with those of precocious puberty. The investigation also included a brain MRI scan, which revealed a small nodule with regular limits in the left hypothalamic region/tuber cinereum. GnRHa treatment and neurosurgical follow-ups were initiated promptly. The patient showed a reversal of secondary sexual characteristics and stable hamartoma size. This case illustrates the importance of brain MRI scans as part of the assessment of suspected precocious puberty because clinical features do not identify patients with an underlying pathology.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10040029/

Record 519

pmcid: PMC9982112

pmid: 36875449

doi: 10.3389/fendo.2023.1134977

title: Impact of 6-month triptorelin formulation on predicted adult height and basal gonadotropin levels in patients with central precocious puberty.

authors

- Yoo E
- Kim S
- Jung HL
- Shim JY
- Shim JW
- Kim DS
- Kwak JH
- Kim ES
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journal: Frontiers in endocrinology

journalAbbr: Front Endocrinol (Lausanne)

year: 2023

pubdate: 2023

volume: 14

issue:

pages: 1134977

abstract: BACKGROUND: Triptorelin, a long-acting gonadotropin-releasing hormone (GnRH) agonist, is available in 1-, 3-, and 6-month formulations to treat central precocious puberty (CPP). The triptorelin pamoate 22.5-mg 6-month formulation recently approved for CPP offers greater convenience to children by reducing the injection frequency. However, worldwide research on using the 6-month formulation to treat CPP is scarce. This study aimed to determine the impact of the 6-month formulation on predicted adult height (PAH), changes in gonadotropin levels, and related variables. METHODS: We included 42 patients (33 girls and nine boys) with idiopathic CPP treated with a 6-month triptorelin (6-mo TP) formulation for over 12 months. Auxological parameters, including chronological age, bone age, height (cm and standard deviation score [SDS]), weight (kg and SDS), target height (TH), and Tanner stage, were evaluated at baseline, and after 6, 12, and 18 months of treatment. Hormonal parameters, including serum luteinizing hormone (LH), follicle-stimulating hormone (FSH), and estradiol for girls or testosterone for boys, were analyzed concurrently. RESULTS: The mean age at treatment initiation was 8.6 ± 0.83 (8.3 ± 0.62 for girls, 9.6 ± 0.68 for boys). The peak LH level following intravenous GnRH stimulation at diagnosis was 15.47 ± 9.94 IU/L. No progression of the modified Tanner stage was observed during treatment. Compared to baseline, LH, FSH, estradiol, and testosterone were significantly reduced. In particular, the basal LH levels were well suppressed to less than l.0 IU/L, and the LH/FSH ratio was less than 0.66. The bone age/chronological age ratio remained stable with a decreasing trend (1.15 at the start of treatment, 1.13 at 12 months, 1.11 at 18 months). PAH SDS increased during treatment (0.77 ± 0.79 at baseline, 0.87 ± 0.84 at the start of treatment, 1.01 ± 0.93 at six months, and 0.91 ± 0.79 at 12 months). No adverse effects were observed during treatment. CONCLUSION: The 6-mo TP suppressed the pituitary-gonadal axis stably and improved the PAH during treatment. Considering its convenience and effectiveness, a significant shift to long-acting formulations can be expected.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9982112/

Record 520

pmcid: PMC9926372

pmid: 36798680

doi: 10.1177/2050313X231154996

title: Intermammary breast cancer: A rare case of cancer with origin of breast cells in an unusual location.

authors

- Aghajanzadeh M
- Torabi H
- Najafi B
- Talebi P
- Shirini K

journal: SAGE open medical case reports

journalAbbr: SAGE Open Med Case Rep

year: 2023

pubdate: 2023

volume: 11

issue:

pages: 2050313X231154996

abstract: The most common type of cancer among the female population is breast cancer. The most common site for the occurrence of breast cancer is the upper outer quadrant; the upper inner quadrant is the second site, and both the lower outer and the lower inner quadrants are in the third place. This problem is rarely seen in the central portion. Intermammary metastasis due to breast cancer is an infrequent finding. This article presents a 62-year-old lady who presented to the surgical ward with intermammary swelling that appeared suddenly 3 months ago. Ultrasound examination showed a hypoechoic micro-lobulated mass with internal vascularity on the chest wall. Although core needle biopsy suspected invasive ductal carcinoma, both right and left axillary lymph nodes were normal and free. The patient was consulted by an oncologist who recommended radiotherapy before surgery and chemotherapy before and after surgery. This study aims to report and discuss a rare case of intermammary cancer with the origin of breast cells without breast and axillary lymph node involvement. Although the intermammary region is an extremely rare location where breast cancer could occur, its management strategy is the same as other breast cancers.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9926372/

Record 521

pmcid: PMC10546800

pmid: 37795006

doi: 10.4274/ejbh.galenos.2023.2023-5-2

title: Is Breast Imaging in Male Patients With Benign Lumps Necessary? A Retrospective Study to Assess Concordance Between Clinical Diagnosis and Imaging Findings.

authors

- Furtado C
- Stankiewicz A
- Klcova J
- Khan M
- Bajwa S
- Isa ZM

journal: European journal of breast health

journalAbbr: Eur J Breast Health

year: 2023

pubdate: 2023 Oct

volume: 19

issue: 4

pages: 304-310

abstract: OBJECTIVE: Breast imaging for male patients is a controversial topic due to the high prevalence of gynecomastia compared to male breast cancer. Worldwide, men are undergoing more breast imaging despite the low incidence of male breast cancer. Gynecomastia is a benign condition, but the anxiety it causes and unnecessary medical costs are still high. MATERIALS AND METHODS: In accordance with Royal College of Radiology guidelines, a retrospective study was performed in two cycles to determine if mammography or ultrasound should be included in the workup of male patients who were referred to a breast care unit for a lump that was deemed benign by doctors. RESULTS: There was 100% concordance between clinical diagnosis and imaging findings. CONCLUSION: In this population imaging was not necessary in cases of probable gynecomastia and benign conditions found during a clinical assessment. Standardised patient assessment methods can improve care and ensure accurate evaluation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10546800/

Record 522

pmcid: PMC10001215

pmid: 36900795

doi: 10.3390/ijerph20053780

title: Is Preoperative Asymmetry a Predictor of Postoperative Asymmetry in Patients Undergoing Breast Reduction?

authors

- Zawadzki T
- Antoszewski B
- Kasielska-Trojan A

journal: International journal of environmental research and public health

journalAbbr: Int J Environ Res Public Health

year: 2023

pubdate: 2023 Feb 21

volume: 20

issue: 5

pages:

abstract: Plastic surgeons aim to achieve symmetry in breast surgeries, which is the main determinant of chest aesthetics. The aim of this study was to verify if preoperative breast asymmetry is a predictor of postoperative asymmetry in women undergoing breast reduction. In this prospective study, we enrolled 71 women (the mean age 37 years, SD 10 years) with breast hypertrophy who underwent reduction mammaplasty. We collected clinical data including age, height, weight, weight of the resected tissues, and performed pre and postoperative photographic documentation. The following measurements of both breasts were analyzed: volumes (vol), nipple-sternal notch distance (A-sn), difference between nipples' levels (A-A'), nipple-midline distance (A-ml), difference between inframammary folds levels (IF-IF'), distance between inframammary fold and nipple (IF-A), distance between inframammary fold apex and midline (IF-ml). All measurements were performed preoperatively and 6 months after the surgery and asymmetries of all variables were calculated (asy-vol, A-A', asyA-sn, asyA-ml, IF-IF', asyIF-A, asyIF-ml). Postoperative asymmetry of breast volumes and nipples position was not associated with any of the analyzed clinical variables. Postoperative asymmetry of nipples' level was associated with preoperative asymmetry of IF-ml; however, logistic regression did not detect any preoperative measurement influencing postoperative volume and nipples' level asymmetry. Moreover, we found that preoperative asyIF-ml increased the risk of postoperative volume asymmetry, which is above the average (52 cc) (OR = 2.04).

Postoperative breast asymmetry after breast reduction is not related neither to preoperative asymmetries nor clinical variables; however, asymmetry of inframammary fold apex to the midline may be the factor affecting postoperative volumetric asymmetry.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10001215/

Record 523

pmcid: PMC10142323

pmid: 37123806

doi: 10.7759/cureus.36849

title: Journeying Through the Hurdles of Gender-Affirming Care Insurance: A Literature Analysis.

authors

- Patel H
- Camacho JM
- Salehi N
- Garakani R
- Friedman L
- Reid CM

journal: Cureus

journalAbbr: Cureus

year: 2023

pubdate: 2023 Mar

volume: 15

issue: 3

pages: e36849

abstract: Gender-affirming surgery (GAS) has been proven to be successful in the treatment of gender dysphoria. The benefits of providing insurance coverage for transition-related surgeries far surpass the costs of suffering from persistent gender dysphoria, including many positive health outcomes such as decreased rates of substance use, psychiatric illness, and suicide. Despite being deemed a medical necessity, discrepancies in access to treatment and insurance coverage for GAS persist. The purpose of this review is to understand the impact of limited insurance coverage on the well-being of transgender patients. A comprehensive search was conducted utilizing the Preferred Reporting Items for Systematic Reviews and Meta-Analyses (PRISMA) guidelines in SCOPUS and PubMed databases using the terms "insurance" AND "gender affirming surgery." Articles in non-English languages were excluded. Data related to variations in insurance coverage for GAS in the United States were collected. Of the 67 articles reviewed, 29 met the inclusion criteria. When compared to the general population, individuals who identify as transgender have higher rates of being uninsured as of 2020, with only 30 states in the United States providing insurance coverage for transgender and gender non-binary people. Of the 30 states, only 18 provide coverage for GAS, with chondrolaryngoplasty having the highest prevalence of coverage. As evidenced in our review, the persistence of complex insurance regulations impedes transgender individuals' access to equitable care. Overall, this literature review elucidates the variability in insurance coverage as it relates to gender-affirming care. Furthermore, this review highlights the need for additional health policy reforms, in addition to improving physician awareness regarding the hurdles of navigating the insurance world as a transgender patient.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10142323/

Record 524

pmcid: PMC10914104

pmid: 38449551

doi: 10.4103/jwas.jwas_54_23

title: Leydig Cell Testicular Tumor Presenting as Bilateral Breast Masses: A Case Report.

authors

- Ojewola RW
- Aranmolate RA

journal: Journal of the West African College of Surgeons

journalAbbr: J West Afr Coll Surg

year: 2023

pubdate: 2023 Oct-Dec

volume: 13

issue: 4

pages: 119-122

abstract: Leydig cell tumors are rare but are the most common nongerm cell gonadal tumors. They are mostly benign but malignant variants have been reported. Leydig cells constitute the main androgen-synthesizing compartment in adult males but are also capable of estrogen production. This can manifest with clinical features of excessive hormone elaboration. We report a case of a 39-year-old man with abnormal bilateral breast development, reduced libido, and weak erection of 3 years' duration. He never noticed any testicular swelling before presentation. Examination revealed well-developed breasts bilaterally and a mass in the lower pole of the left testis. Scrotal ultrasound confirmed a hypoechoic tumor measuring 2 × 3 cm in the lower pole of the left testis and hormonal evaluation revealed a markedly elevated estradiol level. A diagnosis of estrogen-secreting testicular tumor was made. He had a testis-sparing excision of the scrotal lesion as well as liposuction and excision of glandular tissues of the breasts. He had an uneventful postoperative recovery and was discharged a day after surgery. Histology of excised testicular lesion revealed a benign Leydig cell tumor. Four months following surgery, there was an improvement in libido, erection, and sperm concentration of the patient. The patient was also very satisfied with the cosmetic outcome of the excision of the bilateral gynecomastia. We recommend self-examination of testicles as an important step for early diagnosis of testicular tumors.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10914104/

Record 525

pmcid: PMC10241644

pmid: 37288017

doi: 10.4103/abr.abr_374_21

title: Liposuction versus Periareolar Excision Approach for Gynecomastia Treatment.

authors

- Abdali H
- Rasti M
- Adib Parsa M
- Seyedipour S
- Tavakoli-Fard N

journal: Advanced biomedical research

journalAbbr: Adv Biomed Res

year: 2023

pubdate: 2023

volume: 12

issue:

pages: 93

abstract: BACKGROUND: Gynecomastia (GM) is the increased fibroglandular tissue in the male breast by more than 2 cm, which is palpated under the nipple and areola. An ideal surgical approach aims to reduce the breast size, reach an acceptable breast shape, resect excessive glandular tissue, fatty tissue, and skin fatty tissue and excess skin, relocate the nipple-areolar complex, and avoid scars. Based on its importance, we aimed to compare outcomes of liposuction with and without periareolar incision in patients with GM. MATERIALS AND METHODS: This was a randomized clinical trial on patients referred for plastic surgery. Patients with GM were allocated into two treatment groups. Group A underwent liposuction without any areolar skin incision and group B had liposuction with the areolar skin incision. Patients were followed-up after surgery. Data were analyzed by Statistical Package for the Social Sciences (SPSS) version 20. RESULTS: Sixty patients aged between 20 and 27 years old participated in this study. Three hematomas, two surgical site infections, one nipple hypopigmentation after surgery, and one seroma formation were noted in group B. On the other hand, one hematoma and one seroma formation were noted in group A. The patients in group A were highly satisfied after the liposuction without skin incision procedure compared with group B (P = 0.01). CONCLUSIONS: The management of GM by liposuction, either with the periareolar excision technique or without skin incision, allows the effective removal of fat and glandular tissue of the male breast. Although there was no significant difference regarding postoperation complications between groups, patients' satisfaction should be considered.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10241644/

Record 526

pmcid: PMC10743117

pmid: 38132204

doi: 10.3390/diagnostics13243620

title: Male Breast: A Review of the Literature and Current State of the Art of Diagnostic Imaging Work-Up.

authors

- D'Angelo A
- Portaluri A
- Caprini F
- Sofia C
- Ferrara F
- Condorelli E
- Iaccarino L
- Catanzariti F
- Mancino M
- Trombadori CML
- Belli P
- Marino MA

journal: Diagnostics (Basel, Switzerland)

journalAbbr: Diagnostics (Basel)

year: 2023

pubdate: 2023 Dec 7

volume: 13

issue: 24

pages:

abstract: Pathological conditions affecting the male breast (MB) share some similarities with those found in women, while others are specific to men. The first part of this review provides an overview of MB disorders, exploring the most common types of MB diseases. The second part then emphasizes the state-of-the-art approaches proposed in the literature for screening and follow-up with MB cancer patients, which highlights the importance of tailored strategies for diagnosis, follow-up, and identifying high-risk populations. Considering the increasing attention in recent years on the topic, transgender individuals are also included in this review. Together with the MB, it is an understudied category thus far. This review aims to raise awareness among radiologists that MBs should be approached differently from female breasts, contributing to the advancement of medical knowledge, improving patient outcomes, and promoting early detection of MB disorders. The review also provides an update on breast cancer and screening in the transgender population.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10743117/

Record 527

pmcid: PMC10039315

pmid: 36974228

doi: 10.7759/cureus.35335

title: Mammary Myofibroblastoma in a Male: A Case Report and Review of the Literature.

authors

- Fatani JA
- Alotaibi O
- Jacob M
- Terro K
- Alhajri K

journal: Cureus

journalAbbr: Cureus

year: 2023

pubdate: 2023 Feb

volume: 15

issue: 2

pages: e35335

abstract: Mammary myofibroblastoma is a rare benign tumor. It is mainly seen in older men and postmenopausal women. These tumors can be presented with a palpable mass or can be discovered incidentally on routine screening. A 76-year-old male presented with a palpable breast mass that was increasing in size. The patient underwent wide local excision with no postoperative complications. The pathology finding was consistent with myofibroblastoma. Myofibroblastoma is a rare tumor and should be considered one of the differential diagnoses in breast lumps.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10039315/

Record 528

pmcid: PMC10217528

pmid: 37238291

doi: 10.3390/diagnostics13101807

title: Medical Imaging of Inflammations and Infections of Breast Implants.

authors

- Giovannini E
- Travascio L
- Follacchio GA
- Bauckneht M
- Criscuoli B
- De Cataldo C
- Iozzelli A
- Cimini A
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journal: Diagnostics (Basel, Switzerland)

journalAbbr: Diagnostics (Basel)

year: 2023

pubdate: 2023 May 20

volume: 13

issue: 10

pages:

abstract: Breast implants are widely used for reconstructive and/or cosmetic purposes. Inflammations and infections of breast implants represent important complications in clinical practice. The proper management of complications is necessary: diagnostic imaging plays a key role in detecting sites of inflammation and/or infection. The present review aims to illustrate the radiological findings of these conditions with different imaging techniques, such as mammography (MX), ultrasound (US), magnetic resonance imaging (MRI), and nuclear medicine imaging. A knowledge of these findings is essential for radiologists and nuclear medicine physicians to provide helpful information for the clinical management of these complications.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10217528/

Record 529

pmcid: PMC10113936

pmid: 36200436

doi: 10.17305/bjbms.2022.8175

title: Metabolic aspects of surgical subcutaneous fat removal: An umbrella review and implications for future research.

authors

- Badran S
- Doi SA
- Hamdi M
- Hammouda A
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- Clark J
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- Habib AM

journal: Biomolecules & biomedicine

journalAbbr: Biomol Biomed

year: 2023

pubdate: 2023 Mar 16

volume: 23

issue: 2

pages: 235-247

abstract: Although obesity is a preventable disease, maintaining a normal body weight can be very challenging and difficult, which has led to a significant increase in the demand for surgical subcutaneous fat removal (SSFR) to improve physical appearance. The need for SSFR is further exacerbated because of the global rise in the number of bariatric surgeries, which is currently the single most durable intervention for mitigating obesity. Fat tissue is now recognized as a vital endocrine organ that produces several bioactive proteins. Thus, SSFR-mediated weight (fat) loss can potentially have significant metabolic effects; however, currently, there is no consensus on this issue. This review focuses on the metabolic sequelae after SSFR interventions for dealing with cosmetic body appearance. Data was extracted from existing systematic reviews and the diversity of possible metabolic changes after SSFR are reported along with gaps in the knowledge and future directions for research and practice. We conclude that there is a potential for metabolic sequelae after SSFR interventions and their clinical implications for the safety of the procedures as well as for our understanding of subcutaneous adipose tissue biology and insulin resistance are discussed.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10113936/

Record 530

pmcid: PMC10463894

pmid: 37626398

doi: 10.1186/s12916-023-03032-0

title: Metabolic characteristics and pathogenesis of precocious puberty in girls: the role of perfluorinated compounds.

authors

- Wu J
- Chen J
- Huang R
- Zhu H
- Che L
- Lin Y
- Chang Y
- Shen G
- Feng J

journal: BMC medicine

journalAbbr: BMC Med

year: 2023

pubdate: 2023 Aug 25

volume: 21

issue: 1

pages: 323

abstract: BACKGROUND: Precocious puberty (PP) in girls is traditionally defined as the onset of breast

development before the age of 8 years. The specific biomarkers of premature thelarche (PT) and central precocious puberty (CPP) girls are uncertain, and little is known about their metabolic characteristics driven by perfluorinated compounds (PFCs) and clinical phenotype. This study aimed to screen specific biomarkers of PT and CPP and elucidate their underlying pathogenesis. The relationships of clinical phenotype-serum PFCs-metabolic characteristics were also explored to reveal the relationship between PFCs and the occurrence and development of PT and CPP. METHODS: Nuclear magnetic resonance (NMR)-based cross-metabolomics strategy was performed on serum from 146 PP (including 30 CPP, 40 PT, and 76 unspecified PP) girls and 64 healthy girls (including 36 prepubertal and 28 adolescent). Specific biomarkers were screened by the uni- and multivariate statistical analyses. The relationships between serum PFCs and clinical phenotype were performed by correlation analysis and weighted gene co-expression network analysis to explore the link of clinical phenotype-PFCs-metabolic characteristics in PT and CPP. RESULTS: The disordered trend of pyruvate and butyrate metabolisms (metabolites mapped as formate, ethanol, and 3-hydroxybutyrate) were shared and kept almost consistent in PT and CPP. Eight and eleven specific biomarkers were screened for PT and CPP, respectively. The area under curve of specific biomarker combination was 0.721 in CPP vs. prepubertal, 0.972 in PT vs. prepubertal, 0.646 in CPP vs. prepubertal integrated adolescent, and 0.822 in PT vs. prepubertal integrated adolescent, respectively. Perfluoro-n-heptanoic acid and perfluoro-n-hexanoic acid were statistically different between PT and CPP. Estradiol and prolactin were significantly correlated with PFCs in CPP and PT. Clinical phenotypes and PFCs drive the metabolic characteristics and cause metabolic disturbances in CPP and PT. CONCLUSIONS: The elevation of formate, ethanol, and 3-hydroxybutyrate may serve as the early diagnostic indicator for PP in girls. But the stratification of PP still needs to be further determined based on the specific biomarkers. Specific biomarkers of CPP and PT exhibited good sensitivity and can facilitate the classification diagnosis of CPP and PT. PFC exposure is associated with endocrine homeostasis imbalance. PFC exposure and/or endocrine disturbance directly or indirectly drive metabolic changes and form overall metabolic network perturbations in CPP and PT.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10463894/

Record 531

pmcid: PMC10157993

pmid: 37138289

doi: 10.1186/s12905-023-02369-1

title: Mother's perceptions and concerns over sharing sexual and reproductive health information with their adolescent daughters- A qualitative study among mothers of adolescent girls aged 14-19 years in the developing world, Sri Lanka.

authors

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journal: BMC women's health

journalAbbr: BMC Womens Health

year: 2023

pubdate: 2023 May 3

volume: 23

issue: 1

pages: 223

abstract: INTRODUCTION: Studies across the world have shown that parent-adolescent SRH communication can prevent unhealthy sexual and reproductive health (SRH) practices among adolescents and promote their sexual and reproductive health. Parents have the ability to provide individualized sex education to suit the needs of their children, their families, and societal values. The fact that children have more opportunities in the family, makes parent-based sexuality education of children a better approach to Sri Lankan context. OBJECTIVE: To explore mother's perceptions and concerns over sharing SRH information with their daughters among Sinhalese mothers of adolescent girls aged 14-19 years in Sri Lanka. METHODOLOGY: Six Focus group discussions (FGD) were carried out among mothers of adolescent girls aged 14-19 years. Each focus group discussion included 10-12 participants who were recruited using the purposive sampling method. A focus group discussion guide developed after an extensive literature search and expert opinion was used to retrieve information from mothers. Data management and analysis mainly followed an inductive approach to thematic analysis principles. The findings were presented in narrative form using respondents' quotes in their own words and were developed into codes and themes. RESULTS: The mean age of the participants was 43.5 years while 62.4% (n = 40) were educated above the Ordinary level. Analysis of data identified eight main themes from the FGDs. Many mothers thought that sexual and reproductive information is important to adolescent girls. They tried to keep their girl adolescents informed about Adolescent sexual and reproductive health (ASRH) issues. They preferred abstinence-only education over abstinence-plus education. Lack of skills and lack of knowledge on adolescent SRH issues was a major challenge identified by the mothers to communicate SRH matters with their children. CONCLUSIONS AND RECOMMENDATIONS: Although mothers perceived their role as primary sex educators to their children, they were not confident about their knowledge and skills in discussing SRH matters with children. Implementation of interventions to improve mothers' attitudes and skills in communicating SRH matters with children is recommended.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10157993/

Record 532

pmcid: PMC10253870

pmid: 37297902

doi: 10.3390/jcm12113708

title: Nagor Impleo Round Silicone Gel Breast Implants: Early Outcome Analysis after 340 Primary Breast Augmentations.

authors

- Zaussinger M
- Duscher D
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journal: Journal of clinical medicine

journalAbbr: J Clin Med

year: 2023

pubdate: 2023 May 27

volume: 12

issue: 11

pages:

abstract: For decades, implant-based breast augmentation has been one of the most performed surgical procedures for cosmetic purposes around the world. Hence, novel manufactured implants should be critically investigated to prove them safe and effective. Here, the authors describe the first independently conducted clinical study on Nagor Impleo textured round breast implants. For this retrospective study, outcomes of 340 consecutive female patients undergoing primary cosmetic breast augmentation were analyzed. Demographic

and surgical data as well as outcomes and complications were evaluated. Furthermore, a survey concerning effectiveness and aesthetic satisfaction after breast augmentation was examined. All 680 implants were placed in a submuscular plane with incisions at the inframammary fold. The main indications for surgery were hypoplasia and hypoplasia with asymmetry. The mean implant volume was 390 cc and the main type of projection was high profile. The most common complications were hematoma and capsular contracture (0.9 percent, respectively). The overall revision rate for complications was 2.4%. Additionally, almost all patients showed increased quality of life and aesthetic satisfaction after a breast augmentation. Hence, all patients would undergo breast augmentation again with these newly launched devices. Nagor Impleo implants demonstrate a low complication rate and high safety profile. Although high aesthetic satisfaction and quality of life results were achieved, analysis of an even larger series over a longer period of time would be beneficial to evaluate the reliability of this implant.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10253870/

Record 533

pmcid: PMC10628117

pmid: 37942186

doi: 10.1002/ccr3.8158

title: Necrotizing fasciitis in neonates: A case report and review of literature.

authors

- Oboodi R
- Barzegar H
- Behzadi R

journal: Clinical case reports

journalAbbr: Clin Case Rep

year: 2023

pubdate: 2023 Nov

volume: 11

issue: 11

pages: e8158

abstract: Early recognition and treatment of necrotizing fasciitis reduce mortality rate which is 24.1% among neonates. Antibiotics and debridement are common treatments. Hyperbaric oxygen and negative pressure wound healing show potential but need further investigation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10628117/

Record 534

pmcid: PMC10406013

pmid: 37554865

doi: 10.1097/MS9.0000000000001036

title: Neonatal giant mastauxe: a case report.

authors

- Pant VP
- Mishra D

journal: Annals of medicine and surgery (2012)

journalAbbr: Ann Med Surg (Lond)

year: 2023

pubdate: 2023 Aug

volume: 85

issue: 8

pages: 4134-4136

abstract: UNLABELLED: Giant mastauxe is an uncomplicated physiological enlargement of the breasts in newborns, characterized by a breast bud diameter greater than 3 cm. While most newborns have some degree of breast enlargement, giant mastauxe is rare, which may be unnecessarily intervened upon. CASE PRESENTATION: A 13-day-term male neonate presented with bilateral breast enlargement and milky nipple discharge, which worsened upon breast massage. The mother had an uneventful pregnancy, and neither the mother nor the baby had a history of drug intake. The examination revealed soft, non-tender, non-erythematous bilateral breast swellings measuring 3.6×3.8 cm on the right side and 3.2×3.4 cm on the left side. Ultrasonography confirmed their hypoechoic nature with minimum vascularization. The white blood cell count, C-reactive protein levels, and culture of nipple discharge yielded normal results. The baby was discharged, and upon follow-up after 2 weeks, it was observed that breast swellings had spontaneously reduced in size. DISCUSSION: Neonatal breast enlargement can occur as a result of either the transplacental transfer of maternal estrogens or declining levels of estrogens toward the end of pregnancy, leading to hyperprolactinemia. However, the exact reasons behind the occurrence of giant mastauxe in some newborns remain incompletely understood. It may be due to increased sensitivity of the breast tissue to estrogens. CONCLUSION: Proper clinical examination, along with the use of ultrasonography and/or laboratory investigations if required, is essential to differentiate giant mastauxe from neonatal mastitis or breast abscess. Treatment primarily involves observation and providing reassurance to parents.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10406013/

Record 535

pmcid: PMC10683562

pmid: 38018575

doi: 10.1002/iid3.1100

title: New insights on the potential effect of progesterone in Covid-19: Anti-inflammatory and immunosuppressive effects.

authors

- Al-Kuraishy HM
- Al-Maiahy TJ
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journal: Immunity, inflammation and disease

journalAbbr: Immun Inflamm Dis

year: 2023

pubdate: 2023 Nov

volume: 11

issue: 11

pages: e1100

abstract: BACKGROUND: Coronavirus disease 2019 (COVID-19) is a pandemic disease caused by severe acute respiratory syndrome CoV type 2 (SARS-CoV-2). COVID-19 is higher in men than women and sex hormones have immune-modulator effects during different viral infections, including SARS-CoV-2 infection. One of the essential sex hormones is progesterone (P4). AIMS: This review aimed to reveal the association between P4 and Covid-19. RESULTS AND DISCUSSION: The possible role of P4 in COVID-19 could be beneficial through the modulation of inflammatory signaling pathways, induction of the release of anti-inflammatory cytokines, and inhibition release of pro-inflammatory cytokines. P4 stimulates skew of naïve T cells from inflammatory Th1 toward anti-inflammatory Th2 with activation release of anti-inflammatory cytokines, and activation of regulatory T cells (Treg) with decreased interferon-gamma production that increased during SARS-CoV-2 infection. In addition, P4 is regarded as a potent antagonist of mineralocorticoid receptor (MR), it could reduce MRs that were activated by stimulated aldosterone from high AngII during SARS-CoV-2. P4 active metabolite allopregnanolone is regarded as a neurosteroid that acts as a positive modulator of γ-aminobutyric acid (GABA A) so it may reduce neuropsychiatric manifestations and dysautonomia in COVID-19 patients. CONCLUSION: Taken together, the anti-inflammatory and immunomodulatory properties of P4 may improve central and peripheral complications in COVID-19.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10683562/

Record 536

pmcid: PMC10919733

pmid: 38455622

doi: 10.52225/narra.v3i3.205

title: Normal intellectual ability and hyperprolactinemia as unique clinical manifestations of congenital hypothyroidism: A case report and review of hypotheses.

authors

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- Debbyousha M
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journal: Narra J

journalAbbr: Narra J

year: 2023

pubdate: 2023 Dec

volume: 3

issue: 3

pages: e205

abstract: Congenital hypothyroidism is the deficiency of thyroid hormone in infants and hyperprolactinemia is frequently observed. Previously reported cases typically involve intellectual disability, highlighting this particular unique case report to the first reported patient demonstrating normal intellectual ability despite experiencing growth and gonad dysfunction. This study aims to present a case and review medical hypotheses related to the patient's condition. A 19-year-old female presented with a chief complaint of irregular menstruation for up to 40 days or not occurring at all. The patient experienced the first menstruation at the age of 16 years old. The patient's height was 133 cm, body weight 40 kg, and body mass index 22.61 kg/m2; other family members were normal. Physical examination showed no abnormalities, and laboratory examination showed suppressed serum free T4 (FT4) level (6.41 pmol/L), elevated thyroid stimulating hormone (TSH) level (333.700 µIU/mL), and

elevated prolactin hormone level (32.03 ng/mL). Ultrasound of the thyroid gland found hypoplasia of the left and right thyroid glands. The patient was a college student enrolled in a public national university and had never complained about academic performance throughout the patient's education. The patient was diagnosed with congenital hypothyroidism and hyperprolactinemia. The patient was administered up to 100 μg daily of oral levothyroxine, which improved the patient's menstrual cycles. The patient's delayed diagnosis may be attributed to central congenital hypothyroidism being underdiagnosed. We hypothesized that thyroid-releasing hormone receptor (TRHR) gene mutation might contribute to the underlying cause of hyperprolactinemia and normal intellectual ability of the patient. Further study on the significance of TRHR gene mutations in congenital hypothyroidism is required to improve diagnosis and treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10919733/

Record 537

pmcid: PMC10303205

pmid: 37375604

doi: 10.3390/nu15122701

title: Nutritional Composition of Plant Protein Beverages on China's Online Market: A Cross-Sectional Analysis.

authors

- Zhang J
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- Ji W

journal: Nutrients

journalAbbr: Nutrients

year: 2023

pubdate: 2023 Jun 9

volume: 15

issue: 12

pages:

abstract: Plant protein beverages are gaining popularity due to various reasons such as lactose intolerance, veganism and health claims. This study aimed to conduct a cross-sectional analysis of plant protein beverages sold online in China, with a focus on assessing their nutritional composition. A total of 251 kinds of plant protein beverages were analyzed, including coconut (n = 58), soy (n = 52), oats (n = 49), walnut (n = 14), almond (n = 11), peanut (n = 5), rice (n = 4), other beans (n = 5), mixed nuts (n = 5) and mixed beverages (n = 48), according to the nutrition label on the commercial package and retailer websites. The results showed that, except for soy beverages, plant protein beverages generally had low protein content, cereal beverages showed relatively high energy and carbohydrate levels, and all plant protein beverages had low sodium content. Additionally, the fortification rate of vitamins and minerals in the analyzed plant protein beverages was found to be extremely low, at only 13.1%. Given the substantial variation in the nutritional composition of plant protein beverages, consumers should pay more attention to the nutrition facts and ingredient information when choosing these beverages.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10303205/

Record 538

pmcid: PMC10032490

pmid: 36947549

doi: 10.1371/journal.pone.0283510

title: Ongoing increasing trends in central precocious puberty incidence among Korean boys and girls from 2008

to 2020.

authors

- Kang S
- Park MJ
- Kim JM
- Yuk JS
- Kim SH

journal: PloS one

journalAbbr: PLoS One

year: 2023

pubdate: 2023

volume: 18

issue: 3

pages: e0283510

abstract: BACKGROUND: Over the last few decades, there has been growing evidence of earlier onset and progression of puberty worldwide. This population-based longitudinal cohort study aimed to analyze the change in the annual incidence rate of central precocious puberty (CPP) among Korean children over the most recent decade, using the national registry data. METHOD: The International Statistical Classification of Diseases and Related Health Problems, Tenth Revision (ICD-10) and insurance claims for gonadotropin-releasing hormone agonist (GnRHa) treatment were used to identify CPP patients who were using the Korean Health Insurance Review & Assessment Service (HIRA) database between 2008 and 2020. Patients who began GnRHa therapy before the age of 9 and 10 for girls and boys, respectively, were included in the study. RESULTS: A total of 6,906 boys and 126,377 girls were diagnosed with CPP between 2008 and 2020. The annual incidence of CPP increased by 83.3 times in boys (from 1.2 to 100 per 100,000 persons) and by 15.9 times in girls (from 88.9 to 1414.7 per 100,000 persons). The age-specific annual incidence of CPP increased remarkably more in older children than in younger ones; the 2020 CPP incidence among 9-year-old boys and 8-year-old girls reached 705.2 and 7,967.3 per 100,000 persons, respectively. The annual prevalence of CPP in boys and girls increased from 2.7 to 206.5 (76.5 times) and from 141.8 to 3439.9 (24.3 times) per 100,000 persons, respectively. CONCLUSION: Based on GnRHa treatment insurance claims, our study suggests that the annual incidence of CPP has substantially increased in Korea during the past 13 years. These findings highlight the importance of meticulous judgment by doctors in determining GnRHa treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10032490/

Record 539

pmcid: PMC10377960

pmid: 37508611

doi: 10.3390/children10071114

title: Ovarian Masses in Children and Adolescents: A Review of the Literature with Emphasis on the Diagnostic Approach.

authors

- Birbas E
- Kanavos T
- Gkrozou F
- Skentou C
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- Vatopoulou A

journal: Children (Basel, Switzerland)

journalAbbr: Children (Basel)

year: 2023

pubdate: 2023 Jun 27

volume: 10

issue: 7

pages:

abstract: Most abdominal masses in the pediatric population derive from the ovaries. Ovarian masses can occur in all ages, although their incidence, clinical presentation and histological distribution vary among different age groups. Children and adolescents may develop non-neoplastic ovarian lesions, such as functional cysts, endometrioma, torsion, abscess and lymphangioma as well as neoplasms, which are divided into germ cell, epithelial, sex-cord stromal and miscellaneous tumors. Germ cell tumors account for the majority of ovarian neoplasms in the pediatric population, while adults most frequently present with epithelial tumors. Mature teratoma is the most common ovarian neoplasm in children and adolescents, whereas dysgerminoma constitutes the most frequent ovarian malignancy. Clinical manifestations generally include abdominal pain, palpable mass, nausea/vomiting and endocrine alterations, such as menstrual abnormalities, precocious puberty and virilization. During the investigation of pediatric ovarian masses, the most important objective is to evaluate the likelihood of malignancy since the management of benign and malignant lesions is fundamentally different. The presence of solid components, large size and heterogenous appearance on transabdominal ultrasonography, magnetic resonance imaging and computed tomography indicate an increased risk of malignancy. Useful tumor markers that raise concern for ovarian cancer in children and adolescents include alpha-fetoprotein, lactate dehydrogenase, beta subunit of human chorionic gonadotropin, cancer antigen 125 and inhibin. However, their serum levels can neither confirm nor exclude malignancy. Management of pediatric ovarian masses needs to be curative and, when feasible, function-preserving and minimally invasive. Children and adolescents with an ovarian mass should be treated in specialized centers to avoid unnecessary oophorectomies and ensure the best possible outcome.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10377960/

Record 540

pmcid: PMC10058027

pmid: 36982560

doi: 10.3390/ijms24065486

title: Overview of BPH: Symptom Relief with Dietary Polyphenols, Vitamins and Phytochemicals by Nutraceutical Supplements with Implications to the Prostate Microbiome.

authors

- Stewart KL
- Lephart ED

journal: International journal of molecular sciences

journalAbbr: Int J Mol Sci

year: 2023

pubdate: 2023 Mar 13

volume: 24

issue: 6

pages:

abstract: Benign prostatic hyperplasia (BPH) is an age-related disorder, which is one of the most prevalent and costly benign neoplasms in men with over 94 million cases worldwide. Starting before or around 50 years of age, there is a linear increase in prostate volume and BPH symptoms, which are influenced by changes in hormonal, inflammatory, growth factors, cell receptor signaling, diet, physical activity, and the microbiome of the prostate that leads to cellular proliferation. While current pharmaceutical or surgical treatments are currently available, each treatment has serious side effects. This dilemma has motived men to seek treatment without negative side effects from medicinal plants such as botanicals, phytochemicals, and vitamins that have established safety records. This narrative overview focuses on several botanicals, phytochemicals and vitamins that are widely used in the treatment of BPH and emphasizes how, in some cases, combinations of these natural ingredients may provide better BPH symptom relief compared to utilization of a single medicinal plant product (monotherapy). Finally, this overview highlights in vitro, in vivo animal studies and mainly clinical data of journal reports published in the past 5 years from January 2018 to January 2023 on BPH and nutraceuticals. Notably, there is an evolving perspective or rethinking of the role that medicinal phytochemicals and natural vitamins usage play; that is, they may hold promise or are likely to alleviate BPH symptoms.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10058027/

Record 541

pmcid: PMC9850652

pmid: 36684480

doi: 10.5662/wjm.v13.i1.1

title: Pandemic and precocious puberty - a Google trends study.

authors

- Tselebis A
- Zabuliene L
- Milionis C
- Ilias I

journal: World journal of methodology

journalAbbr: World J Methodol

year: 2023

pubdate: 2023 Jan 20

volume: 13

issue: 1

pages: 1-9

abstract: BACKGROUND: Recent publications from several countries have reported that more young people (mainly girls) are experiencing precocious puberty (PP)/menarche during the coronavirus disease 2019 pandemic compared to the past. This variation is attributed to the stress of confinement, lack of exercise, obesity and disturbed sleep patterns. A common feature of the relevant papers, however, is the small number of reported cases of PP. Studies have shown that searches for diseases on the internet also reflect to some extent the epidemiology of these diseases. AIM: To estimate, through internet searches for PP, any changes in the epidemiology of PP. METHODS: We assessed in Google Trends searches for 21 PP-related terms in English internationally (which practically dwarf searches in other languages), in the years 2017-2021. Additionally, we assessed local searches for selected terms, in English and local languages, in countries where a rise in PP has been reported. Searches were collected in Relative Search Volumes format and analyzed using Kendall's Tau test, with a statistical significance threshold of $P < 0.05$. RESULTS: Internationally, searches for three PP-related terms showed no noticeable change over the study period, while searches for eight terms showed a decrease. An increase was found over time in searches for nine PP-related terms. Of the 17 searches in English and local languages, in countries where a rise in PP has been reported, 5 showed a significant increase over time. CONCLUSION: Over the study period, more than half of the search terms showed little change or

declined. The discrepancy between internet searches for PP and the reported increase in the literature is striking. It would be expected that a true increase in the incidence of PP would also be aptly reflected in Google trends. If our findings are valid, the literature may have been biased. The known secular trend of decreasing age of puberty may also have played a role.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9850652/

Record 542

pmcid: PMC10034074

pmid: 36969285

doi: 10.3389/fped.2023.1056035

title: Pattern of linear growth and progression of bone maturation for girls with early-onset puberty: A mixed longitudinal study.

authors

- Liu S
- Su Z
- Pan L
- Chen J
- Zhao X
- Wang L
- Zhang L
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journal: Frontiers in pediatrics

journalAbbr: Front Pediatr

year: 2023

pubdate: 2023

volume: 11

issue:

pages: 1056035

abstract: BACKGROUND AND OBJECTIVE: With a worldwide trend to earlier age of onset of puberty, the prevalence of early-onset puberty (EP) among girls has increased. The impact of EP on the pattern of linear growth and bone maturation is unclear. Accordingly, the objective of our study was to describe this pattern for girls with EP in Shenzhen, China. METHODS: A total of 498 untreated girls diagnosed with EP at Shenzhen Children's Hospital, China, between January 2016 and December 2021. A total of 1,307 anthropometric measurements and 1,307 left-hand radiographs were available for analysis. Artificial intelligence (AI) was used to determine bone age (BA). Participants were classified into groups according to chronological age (CA) and BA. The pattern of linear growth (height) and progression of bone maturation was described between groups using the Lambda-Mu-Sigma (LMS) method. Published height-for-CA and height-for-BA norm references for a healthy Chinese population were used for age-appropriate comparisons. RESULTS: The mean CA of appearance of first pubertal signs (breast buds) was 8.1 ± 0.5 years. Compared to norm-referenced data, girls with EP were significantly taller at a CA of 7-10 years. This was followed by a slowing in linear growth after a CA of 10 years, with 71 girls with EP having already achieved their target adult height. From 7 to 10 years of BA, the linear growth was slower in the EP group compared to norm-reference values. This was followed by a period of catch-up growth at 11.2 years of BA, with growth curves approaching norm-referenced values. The BA progressed rapidly from 7 to 8 years of age in about half of the girls with EP (median ΔBA/ΔCA >1.9), slowing, thereafter, until the period of catch-up growth at 11.2 years of BA. CONCLUSIONS: BA provides a more reliable reference than CA to assess growth parameters among girls with EP. Our limited data set does indicate that EP

does not negatively impact final adult height. Therefore, the growth curves from our study are relevant, providing a reference for pediatricians in this clinical population and, thus, preventing over-treatment for EP.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10034074/

Record 543

pmcid: PMC9839820

pmid: 36050608

doi: 10.1007/s00464-022-09550-x

title: Pediatric endoscopic subcutaneous mastectomy (pesma) with liposuction in adolescents with gynecomastia.

authors

- Varlet F
- Esposito C
- Scalabre A
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- Vermersch S
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journal: Surgical endoscopy

journalAbbr: Surg Endosc

year: 2023

pubdate: 2023 Jan

volume: 37

issue: 1

pages: 766-773

abstract: BACKGROUND: Surgical techniques for treatment of gynecomastia are increasingly less invasive. We described technical standardization of pediatric endoscopic subcutaneous mastectomy (PESMA) with liposuction. METHODS: All adolescents with primary gynecomastia, operated using PESMA with liposuction over the period June 2014-July 2021, were included. The video recording of procedures was analyzed to standardize the operative technique. After patient installation, 3 trocars were placed on the mid-axillary line. The technique included 5 steps: (1) subcutaneous injection of lipolysis solution and liposuction; (2) creation of working space using an inflated balloon; (3) gland dissection using 5-mm sealing device; (4) specimen extraction through the largest trocar orifice; and (5) placement of suction drainage tube. RESULTS: Twenty-four male adolescents, operated for Simon's grade 2B and 3 gynecomastia using PESMA with liposuction over the study period, were included. Mean patient age was 16 years (range 15-18). Gynecomastia was bilateral in 19/24 (79.2%) and unilateral in 5/24 (20.8%). One (4.1%) conversion to open was reported. The mean operative time was 87 min (range 98-160) for unilateral and 160 min (range 140-250) for bilateral procedure. The mean length of stay was 2.2 days (range 1-4). Patients wore a thoracic belt for 15 up to 30 days postoperatively. Post-operative complications occurred in 5/24 (20.8%): 2- or 3 mm second-degree burns in 4 (16.7%) and subcutaneous seroma in 1 (4.1%). All complications were Clavien 2 grade and did not require further treatment. Aesthetic outcomes were very good in 21/24 (87.5%). Three (12.5%) boys had persistent minimal breast asymmetry but did never perceive it negatively. CONCLUSION: PESMA combined with liposuction was feasible and safe for surgical treatment of gynecomastia in this selected cohort of patients. Although challenging, this procedure provided good aesthetic results, with no scars on the anterior thoracic wall. Standardization of the operative technique was a key point for successful outcome.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9839820/

Record 544

pmcid: PMC10292588

pmid: 37377831

doi: 10.18295/squmj.9.2022.053

title: Penile Girth Enhancement Using Amniotic Membrane in a Rabbit Model: A stereological study.

authors

- Ariafar A
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- Eslahi A
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journal: Sultan Qaboos University medical journal

journalAbbr: Sultan Qaboos Univ Med J

year: 2023

pubdate: 2023 May

volume: 23

issue: 2

pages: 182-189

abstract: OBJECTIVES: This study aimed to evaluate the efficacy of penile girth enhancement (PGE) using amniotic membrane (AM) as a graft in a rabbit model. Additionally, quantitative histological data of the structure of the penis were obtained by stereological studies. METHODS: This study was conducted at the Histomorphometry and Stereology Research Center, Shiraz University of Medical Sciences, Shiraz, Iran. In this study, 20 adult male rabbits of similar age and weight were allocated to two groups: sham surgery and surgery+AM. Both groups underwent surgery in which a longitudinal I-shaped midline incision was made in the tunica albuginea on the dorsal surface of the penis. The surgery+AM group underwent PGE using AM as a graft. The penile length and mid circumference were measured using a vernier caliper before and two months after the surgery. RESULTS: The mean total volume and diameter of the penis significantly increased in the surgery+AM group (P <0.03 and P <0.04, respectively). On stereological evaluation, a significant increase in the mean volumes of the tunica albuginea and corpora cavernosa was observed in the surgery+AM group compared to the sham group (P <0.01 and P <0.03, respectively). Additionally, the mean volume densities of the collagen bundles, muscle fibres, cavernous sinuses, and the total number of fibroblasts and smooth muscle cells increased in the surgery+AM group compared to the sham group (P <0.05 each). No infections, bleeding or other complications were observed. CONCLUSION: The use of AM as a graft is a method that shows promising results for material use in penile enhancement. Thus, it may be considered for PGE in the future.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10292588/

Record 545

pmcid: PMC10628410

pmid: 37942036

doi: 10.21037/acr-23-101

title: Perioperative nursing care speed up recover of a male patient after bilateral endoscopic mastectomy: a case report.

authors

- Wang LP
- Salzberg CA
- Benson JR

- Rocco N
- Liang SS

journal: AME case reports

journalAbbr: AME Case Rep

year: 2023

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volume: 7

issue:

pages: 48

abstract: BACKGROUND: Gynecomastia (GYN) is mainly caused by the benign proliferation of mammary glands and adipose tissue. Currently, up to 38% of adult males have GYN. GYN can be caused by systemic diseases, obesity, endocrine disorders (such as liver cirrhosis, which impairs estrogen inactivation), malignant tumors, and medications. Surgical intervention is required after 12 months pharmacological treatment of GYN was no response who have endocrine disorder, or due to psychological and physiological factors, young patients have a higher demand for surgical intervention. Recent advances in minimally invasive endoscopic surgery, with the advantage of rapid rehabilitation, have markedly improved the surgical management of GYN. CASE DESCRIPTION: In November 2021, we admitted a young patient with bilateral GYN whose problem began several years prior and for which he sought surgical intervention. After comprehensive evaluation and psychological consultation, he underwent surgical treatment. The present case report summarizes our experience in nursing this patient. CONCLUSIONS: Perioperative nursing care is essential in the management of patients undergoing endoscopic surgical treatment for GYN. The nursing team must be knowledgeable about the procedure, assess and manage the patient's pain, monitor vital signs, prevent infections, and provide emotional support to the patient. With proper nursing care, patients can recover smoothly and quickly after endoscopic surgical treatment of GYN.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10628410/

Record 546

pmcid: PMC10576221

pmid: 37842397

doi: 10.7759/cureus.45206

title: Phyllodes Tumor of the Breast Complicated With Mastitis.

authors

- Das SS
- Mestha A
- Navlani S
- Hasan Ghazi Mohd E

journal: Cureus

journalAbbr: Cureus

year: 2023

pubdate: 2023 Sep

volume: 15

issue: 9

pages: e45206

abstract: One of the rarest fibro-epithelial neoplasms of the breast during pregnancy is the phyllodes tumor (PT).

It is typically a painless, bi-phasic, and rapidly growing neoplasm that resembles fibroadenomas. It is still unclear if the neoplasm is hormone-dependent during pregnancy. It is often challenging to diagnose and treat PT. Herein, we report a case of a 30-year-old female at 31 weeks gestation who was diagnosed with a benign phyllodes tumor of her breast with concurrent mastitis. She was first seen during her third trimester where the neoplasm was around 5 cm as reported by the ultrasound (US) examination. Her biopsy report was suggestive of a PT and she was advised surgery with excision of the tumor margin, but she refused. Ten days after her delivery she presented to the emergency department with a fever and a hard, engorged, erythematous, and tender left breast. She was diagnosed with mastitis of the left breast. She then underwent incision and drainage of the left breast that drained purulent milk; additionally, large necrotic grape-like tissues were removed and were confirmed by the histopathology report as a benign phyllodes tumor of the breast.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10576221/

Record 547

pmcid: PMC9864699

pmid: 36678189

doi: 10.3390/nu15020317

title: Phytoestrogens and Health Effects.

authors

- Canivenc-Lavier MC
- Bennetau-Pelissero C

journal: Nutrients

journalAbbr: Nutrients

year: 2023

pubdate: 2023 Jan 9

volume: 15

issue: 2

pages:

abstract: Phytoestrogens are literally estrogenic substances of plant origin. Although these substances are useful for plants in many aspects, their estrogenic properties are essentially relevant to their predators. As such, phytoestrogens can be considered to be substances potentially dedicated to plant-predator interaction. Therefore, it is not surprising to note that the word phytoestrogen comes from the early discovery of estrogenic effects in grazing animals and humans. Here, several compounds whose activities have been discovered at nutritional concentrations in animals and humans are examined. The substances analyzed belong to several chemical families, i.e., the flavanones, the coumestans, the resorcylic acid lactones, the isoflavones, and the enterolignans. Following their definition and the evocation of their role in plants, their metabolic transformations and bioavailabilities are discussed. A point is then made regarding their health effects, which can either be beneficial or adverse depending on the subject studied, the sex, the age, and the physiological status. Toxicological information is given based on official data. The effects are first presented in humans. Animal models are evoked when no data are available in humans. The effects are presented with a constant reference to doses and plausible exposure.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9864699/

Record 548

pmcid: PMC10652332

pmid: 38024650

doi: 10.1210/jendso/bvad135

title: Precocious Puberty in Hypothyroidism: Mini-Review of Van Wyk-Grumbach Syndrome.

authors

- Fernandez-Gonzalez SM
- Perez Vila MM
- Prado-Carro AM

journal: Journal of the Endocrine Society

journalAbbr: J Endocr Soc

year: 2023

pubdate: 2023 Nov 2

volume: 7

issue: 12

pages: bvad135

abstract: Severe hypothyroidism can affect a variety of organs and can develop atypical manifestations. Peripheral precocious puberty may be secondary to other endocrinological diseases, which must be taken into account in the differential diagnosis in order to avoid unnecessary additional tests. Van Wyk-Grumbach syndrome is an infrequent manifestation characterized by severe hypothyroidism and incomplete precocious puberty. Diagnosis is made by clinical and complementary tests, and the main treatment goal is to achieve euthyroidism through hormone replacement. Prognosis is good once the treatment is established. The aim of this study is to review the available literature about Van Wyk-Grumbach syndrome following the PRISMA statement, and to present the first clinical case published in Spain. We have included the articles published during the period from 1905 to week 40 of 2022. A total of 68 articles have been selected for study and analysis, within which there are 99 published clinical cases. Girls accounted for 92.1% of cases (median age at the diagnosis 8.5 years). Metrorrhagia was the most prevalent symptom, present in 80.5% of the girls. Abdominal ultrasound was performed in 93.3% of the girls and 97.8% of them had at least one ovarian cyst. All cases were treated with levothyroxine, responding satisfactorily after the first doses of treatment. To conclude, Van Wyk-Grumbach syndrome is characterized by severe hypothyroidism and incomplete precocious puberty, which is important to keep in mind in order to avoid complementary exams and unnecessary surgical interventions.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10652332/

Record 549

pmcid: PMC10663169

pmid: 38021712

doi: 10.7759/cureus.47485

title: Precocious Puberty: Types, Pathogenesis and Updated Management.

authors

- Alghamdi A

journal: Cureus

journalAbbr: Cureus

year: 2023

pubdate: 2023 Oct

volume: 15

issue: 10

pages: e47485

abstract: Precocious puberty (PP) means the appearance of secondary sexual characters before the age of eight years in girls and nine years in boys. Puberty is indicated in girls by the enlargement of the breasts (thelarche) in girls and in boys by the enlargement of the testes in either volume or length (testicular volume = 4

mL, testicular length = 25 mm, or both). Two types of PP are recognized - namely central PP (CPP) and peripheral PP (PPP). This paper aims to describe the clinical findings and laboratory workup of PP and to illustrate the new trends in the management of precocious sexual maturation. Gonadotropin-releasing hormone (GnRH)-independent type (PPP) refers to the development of early pubertal maturation not related to the central activation of the hypothalamic-pituitary-gonadal (HPG) axis. It is classified into genetic or acquired disorders. The most common forms of congenital or genetic causes involve McCune-Albright syndrome (MAS), familial male-limited PP, and congenital adrenal hyperplasia. The acquired causes include exogenous exposure to androgens, functioning tumors or cysts, and the pseudo-PP of profound primary hypothyroidism. On the other hand, CPP is the most common and it is a gonadotropin-dependent form. It is due to premature maturation of the HPG axis. CPP may occur as genetic alterations, such as MKRN3, DLK1, or KISS1;as a part of mutations in the epigenetic factors that regulate the HPG axis, such as Lin28b and let-7; or as a part of syndromes, central lesions such as hypothalamic hamartoma, and others. A full, detailed history and physical examination should be taken. Furthermore, several investigations should be conducted for both types of PP, including the estimation of serum gonadotropins such as luteinizing and follicle-stimulating hormones and sex steroids, in addition to a radiographic workup and thyroid function tests. Treatment depends on the type of PP: Long-acting GnRHa, either intramuscularly or implanted, is the norm of care for CPP management, while in PPP, especially in congenital adrenal hyperplasia, the goal of management is to suppress adrenal androgen secretion by glucocorticoids. In addition, anastrozole and letrozole - third-generation aromatase inhibitors - are more potent for MAS.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10663169/

Record 550

pmcid: PMC10637039

pmid: 37243694

doi: 10.1093/jnci/djad101

title: Prediction of breast cancer risk for sisters of women attending screening.

authors

- Mao X
- He W
- Eriksson M
- Lindström LS
- Holowko N
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- Hammarström M
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- Humphreys K
- Easton D
- Hall P
- Czene K

journal: Journal of the National Cancer Institute

journalAbbr: J Natl Cancer Inst

year: 2023

pubdate: 2023 Nov 8

volume: 115

issue: 11

pages: 1310-1317

abstract: BACKGROUND: Risk assessment is important for breast cancer prevention and early detection. We aimed to examine whether common risk factors, mammographic features, and breast cancer risk prediction scores of a woman were associated with breast cancer risk for her sisters. METHODS: We included 53 051 women from the Karolinska Mammography Project for Risk Prediction of Breast Cancer (KARMA) study. Established risk factors were derived using self-reported questionnaires, mammograms, and single nucleotide polymorphism genotyping. Using the Swedish Multi-Generation Register, we identified 32 198 sisters of the KARMA women (including 5352 KARMA participants and 26 846 nonparticipants). Cox models were used to estimate the hazard ratios of breast cancer for both women and their sisters, respectively. RESULTS: A higher breast cancer polygenic risk score, a history of benign breast disease, and higher breast density in women were associated with an increased risk of breast cancer for both women and their sisters. No statistically significant association was observed between breast microcalcifications and masses in women and breast cancer risk for their sisters. Furthermore, higher breast cancer risk scores in women were associated with an increased risk of breast cancer for their sisters. Specifically, the hazard ratios for breast cancer per 1 standard deviation increase in age-adjusted KARMA, Breast and Ovarian Analysis of Disease Incidence and Carrier Estimation Algorithm (BOADICEA), and Tyrer-Cuzick risk scores were 1.16 (95% confidence interval [CI] = 1.07 to 1.27), 1.23 (95% CI = 1.12 to 1.35), and 1.21 (95% CI = 1.11 to 1.32), respectively. CONCLUSION: A woman's breast cancer risk factors are associated with her sister's breast cancer risk. However, the clinical utility of these findings requires further investigation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10637039/

Record 551

pmcid: PMC10729149

pmid: 38070936

doi: 10.1136/bmjopen-2022-069637

title: Prevalence and risk factors of metabolic syndrome in Ethiopia: describing an emerging outbreak in HIV clinics of the sub-Saharan Africa - a cross-sectional study.

authors

- Abdela AA
- Yifter H
- Reja A
- Shewaamare A
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journal: BMJ open

journalAbbr: BMJ Open

year: 2023

pubdate: 2023 Dec 9

volume: 13

issue: 12

pages: e069637

abstract: OBJECTIVES: HIV-induced chronic inflammation, immune activation and combination antiretroviral therapy (cART) are linked with adverse metabolic changes known to cause cardiovascular adversities. This study evaluates the prevalence of lipodystrophy, and metabolic syndrome (MetS), and analyses risk factors in HIV-infected Ethiopians taking cART. METHODS: A multicentre cross-sectional study was conducted at tertiary-level hospitals. Eligible participants attending the HIV clinics were enrolled. Sociodemographic, anthropometric, clinical, HIV treatment variables, lipid profile, fasting blood glucose level, risk factors and components of MetS, also lipodystrophy, were studied. Data were analysed by SPSS statistical package V.25 with descriptive and analytical statistics. For multivariable analysis of risk factors, a logistic regression model was used. Results were

presented in frequency and percentages, mean±SD, or median+IQR. Statistical significance was taken as p<0.05. RESULTS: Among 518 studied participants, two-thirds were females, and the mean age of the study population was 45 years (SD=11). The mean duration of cART was 10 years (SD=4). Median CD4 count was 460 cells/mm 3 . The prevalence of MetS according to the Adult Treatment Panel III (2005) criteria was 37.6%. In multivariable analysis, independent risk factors for MetS were age >45 years (aHR 1.8, 95% CI 1.2 to 2.4), female sex (aHR 1.8, 95% CI 1.1 to 2.8), body mass index (BMI) > 25 kg/m 2 (aHR 2.7, 95% CI 1.8 to 4.1), efavirenz-based cART (aHR 2.8, 95% CI 1.6 to 4.8) and lopinavir/ritonavir-based cART (aHR 3.7, 95% CI 1.0 to 13.3). The prevalence of lipodystrophy was 23.6%. Prior exposure to a stavudine-containing regimen was independently associated with lipodystrophy (aHR 3.1, 95% CI 1.6 to 6.1). CONCLUSION: Our study revealed 38% of the participants had MetS indicating considerable cardiovascular disease (CVD) risks. Independent risk factors for MetS were BMI≥25 kg/m 2 , efavirenz and lopinavir/ritonavir-based cART, female sex and age ≥45 years. In addition to prevention, CVD risk stratification and management will reduce morbidity and mortality in people with HIV infection.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10729149/

Record 552

pmcid: PMC10205444

pmid: 37196473

doi: 10.1016/j.ijscr.2023.108163

title: Primary angiosarcoma of breast in an octogenarian woman: A case report with literature review.

authors

- Vohra LM
- Jabeen D
- Idrees R

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2023

pubdate: 2023 Jun

volume: 107

issue:

pages: 108163

abstract: INTRODUCTION AND IMPORTANCE: Primary breast angiosarcomas are endothelial derived breast sarcomas found in younger to middle age groups. The diagnosis of primary breast angiosarcoma in an octogenarian woman is a rare incidence. CASE PRESENTATION: We report a case of 87-year-old postmenopausal woman presenting with history of lump in her right breast for four months. Ultrasound guided biopsy was performed which confirmed the diagnosis of angiosarcoma hence, subjected to simple mastectomy. She was doing quite well for one year when ultimately, she developed metastatic disease and couldn't survive more. CLINICAL DISCUSSION: Microscopically, these tumours are classified into grades I, II and III. Hematogenous route has been taken for metastasis having lungs being most involved. There are limited case reports and studies that have investigated the use of adjuvant radio/chemotherapy. CONCLUSION: Primary angiosarcoma of breast is a rare disease in old age group with limited treatment options which resulted in poor prognosis and early relapse.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10205444/

Record 553

pmcid: PMC10688996

pmid: 38045429

doi: 10.1159/000533476

title: Primary Anterior Medistinal Choriocarcinoma in Male with Lung Metastasis and Pituitary Microadenoma: A Case Report.

authors

- Liu D
- Ye Q
- Ye M
- Yang Y

journal: Case reports in oncology

journalAbbr: Case Rep Oncol

year: 2023

pubdate: 2023 Jan-Dec

volume: 16

issue: 1

pages: 1518-1527

abstract: The authors report a case of a 29-year-old male presented with bilateral breast enlargement with no significant past medical history or estrogen exposure. Serum β-human chorionic gonadotropin (HCG) was 14,306.60 mIU and positron emission tomography-computed tomography discovered a malignant mass on the right side of anterior superior mediastinum. Magnetic resonance imaging demonstrated pituitary microadenoma. Pathological biopsy showed poorly differential pituitary adenoma and immunohistochemical staining displayed that CK(+), PLAP(-), AFP(-), HCG(+), CD30(-), Oct3/4(-), CK7(+), TTF-1(-), CD117(-), Ki 67(80+), CK5/6(-), EMA(partial+), inhibin(partial+). A diagnosis of primary anterior mediastinal choriocarcinoma metastasis to bilateral lungs accompanied with pituitary microadenoma was confirmed. Then the patient received chemotherapy combined with immunotherapy. But serum β-HCG level was still above the normal, and unfortunately, the patient died 6 months after his diagnosis. This case inspires us to think of the possibility of choriocarcinoma when a man presents gynecomastia or lung metastatic symptoms, adding Opdivo to the chemotherapy might not improve the poor treatment outcomes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10688996/

Record 554

pmcid: PMC10543409

pmid: 37790899

doi: 10.2147/CMAR.S426780

title: Primary Breast Burkitt Lymphoma. A Case Report of a 16-Year-Old Female with a Rapidly Growing Unilateral Breast Mass.

authors

- Mitala Y
- Ssenkumba B
- Nabulya R
- Kitenda FN
- Tumwesigye H
- Birungi A
- Dratu Chau R
- Mushabe B

- Baker N
- Atwine R

journal: Cancer management and research

journalAbbr: Cancer Manag Res

year: 2023

pubdate: 2023

volume: 15

issue:

pages: 1047-1052

abstract: INTRODUCTION: Primary breast Burkitt lymphoma is extremely rare. Commonly endemic Burkitt lymphoma presents with abdominal, jaw, periorbital, or genitourinary mass. CASE PRESENTATION: We report a case of a 16-year-old girl with rapidly enlarging left breast swelling associated with evening fevers. This was later confirmed to be stage 1 primary breast Burkitt lymphoma involving the left breast. This represents the first described case of primary breast endemic Burkitt lymphoma in Uganda. She was started on chemotherapy and exhibited an impressive response to the drugs. CONCLUSION: This case raises awareness of rare sites for endemic Burkitt lymphoma in Uganda. Accurately diagnosing this case was of great importance since it determined the treatment modality (mastectomy or not) which would have an everlasting impact on her life.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10543409/

Record 555

pmcid: PMC10257786

pmid: 37128794

doi: 10.47162/RJME.64.1.09

title: Primary de novo malignant peripheral nerve sheath tumor of the breast mimicking mastitis. Report of a case and review of the literature.

authors

- Val-Bernal JF
- Hermana S
- Hernanz F
- Moreno G

journal: Romanian journal of morphology and embryology = Revue roumaine de morphologie et embryologie

journalAbbr: Rom J Morphol Embryol

year: 2023

pubdate: 2023 Jan-Mar

volume: 64

issue: 1

pages: 73-82

abstract: Malignant peripheral nerve sheath tumor (MPNST) is a rare tumor representing <0.1% of malignant breast tumors. Here, we report a case of sporadic MPNST of the breast in a 45-year-old woman with a fast-evolving painful mass in the right breast associated with edema, redness, and increased local temperature, simulating mastitis. This presentation has not been reported. A review of the literature since 1992 has revealed 15 MPNST cases, including the present one with sufficient data for analysis. All the cases were women. The ages ranged from 16-60 years (mean 40.5 years). Duration of symptoms varied from four days to 15 years (mean 2.7 years). Tumor size ranged from 2.5-30 cm (mean 10.9 cm). 64.3% of tumors were well-

circumscribed. Tumors were graded as grade 1 (7.7%), grade 2 (38.5%), and grade 3 (53.8%). Three (20%) cases showed histopathological peculiarities. Simple mastectomy was performed in four (26.7%) cases, radical mastectomy in six (40%) cases, and simple excision in five (33.3%) cases. There were no metastases in the six cases where axillary dissection was performed. Seven (53.8%) patients received adjuvant therapy, including chemotherapy and⁄or radiotherapy. Local recurrence was observed in two (16.7%) patients. Distant metastasis was detected in one patient (7.7%) affecting the lung 11 months after radical mastectomy. The follow-up was available for 10 patients (mean 2.2 years, median 1 year). One patient (10%) died of the disease. At the last follow-up, 80% of patients were still alive. The main differential diagnosis includes nonpleomorphic spindle cell tumors.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10257786/

Record 556

pmcid: PMC10475705

pmid: 37272246

doi: 10.4048/jbc.2023.26.e21

title: Pseudoangiomatous Stromal Hyperplasia of the Breast in a Female Adolescent Presenting as Bilateral Gigantomastia.

authors

- Woo SH
- Kang H
- Choi WJ
- Kim EK

journal: Journal of breast cancer

journalAbbr: J Breast Cancer

year: 2023

pubdate: 2023 Aug

volume: 26

issue: 4

pages: 391-396

abstract: Pseudoangiomatous stromal hyperplasia (PASH) is a rare idiopathic proliferative mesenchymal breast disease related to hormonal imbalance, and thus extremely rare in children and adolescents. In addition, PASH manifests as a bilateral gigantomastia in some cases with no established cause or treatment. Here, we report a case of a rapidly developed PASH presenting with bilateral gigantomastia in a 14-year-old premenarchial female patient. Considering the patient's age and emotions and the need for nipple-areolar complex repositioning, we performed reduction mammoplasty rather than total mastectomy despite the possibility of recurrence. Although some masses could not be completely removed, no complications, such as infection, wound dehiscence, or hematoma occurred postoperatively. The patient was stable during the 18-month follow-up period, although an evidence of recurrent and residual disease was noted upon ultrasonography.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10475705/

Record 557

pmcid: PMC9901424

pmid: 36778653

doi: 10.1093/jbi/wbac051

title: Pseudoangiomatous Stromal Hyperplasia: Radiologic-Pathologic Correlation.

authors

- Speer ME
- Yoon EC
- Berg WA
- Chang Sen LQ

journal: Journal of breast imaging

journalAbbr: J Breast Imaging

year: 2023

pubdate: 2023 Jan-Feb

volume: 5

issue: 1

pages: 67-72

abstract: Pseudoangiomatous stromal hyperplasia (PASH) is a benign mesenchymal proliferative lesion of the breast. PASH is postulated to be hormonally induced and predominantly occurs in premenopausal women and postmenopausal women on menopausal hormone therapy. Clinical presentation varies from screen-detected lesions to palpable masses. Imaging findings of PASH are nonspecific. The most common mammographic findings are an oval or round circumscribed non-calcified mass or developing asymmetry. On US, PASH is often seen as an oval hypoechoic mass that may be circumscribed and can have an echogenic rim, or, when manifest as mammographic asymmetry, US may show a corresponding non-mass focal area of echogenic tissue. Limited studies have investigated the MRI appearance, with PASH most often manifesting as non-mass enhancement, or, less often, as an oval or irregular mass with persistent kinetics. Histopathologically, PASH can be mistaken for a fibroadenoma or phyllodes tumor and has features overlapping low-grade angiosarcoma. Assessment of radiologic-pathologic concordance is particularly important as PASH is often an incidental finding, adjacent to the targeted lesion at histopathology. Surgical excision or repeat core-needle biopsy is necessary for discordant suspicious cases. After a benign, concordant diagnosis of PASH, the patient may resume routine screening.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9901424/

Record 558

pmcid: PMC9902184

pmid: 35912427

doi: 10.4097/kja.22268

title: Ramped versus sniffing position in the videolaryngoscopy-guided tracheal intubation of morbidly obese patients: a prospective randomized study.

authors

- Lee S
- Jang EA
- Hong M
- Bae HB
- Kim J

journal: Korean journal of anesthesiology

journalAbbr: Korean J Anesthesiol

year: 2023

pubdate: 2023 Feb

volume: 76

issue: 1

pages: 47-55

abstract: BACKGROUND: Ramped positioning is recommended for intubating obese patients undergoing direct laryngoscopy. However, whether the use of the ramped position can provide any benefit in videolaryngoscopy-guided intubation remains unclear. This study assessed intubation time using videolaryngoscopy in morbidly obese patients in the ramped versus sniffing positions. METHODS: This is a prospective randomized study in patients with morbid obesity (n = 82; body mass index [BMI] ≥ 35 kg/m2). Patients were randomly allocated to either the ramped or the standard sniffing position groups. During the induction of general anesthesia, difficulty in mask ventilation was assessed using the Warters scale. Tracheal intubation was performed using a C-MAC® D-Blade videolaryngoscope, and intubation difficulty was assessed using the intubation difficulty scale (IDS). The primary endpoint was the total intubation time calculated as the sum of the laryngoscopy and tube insertion times. RESULTS: The percentage of difficult mask ventilation (Warters scale ≥ 4) was significantly lower in the ramped (n = 40) than in the sniffing group (n = 41) (2.5% vs. 34.1%, P < 0.001). The percentage of easy intubation (IDS = 0) was significantly higher in the ramped than in the sniffing group (70.0% vs. 7.3%, P < 0.001). The total intubation time was significantly shorter in the ramped than in the sniffing group (22.5 ± 6.2 vs. 40.9 ± 9.0, P < 0.001). CONCLUSIONS: Compared with the sniffing position, the ramped position reduced intubation time in morbidly obese patients and effectively facilitated both mask ventilation and tracheal intubation using videolaryngoscopy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9902184/

Record 559

pmcid: PMC9872972

pmid: 36699203

doi: 10.1097/GOX.0000000000004571

title: Recurrent Extreme Bilateral Gigantomastia Caused by Pseudoangiomatous Stromal Hyperplasia (PASH) Syndrome: A Case Report.

authors

- Parisi P
- Vinci F
- Di Meglio F
- Marruzzo G
- De Luca A
- Ascione A
- Vittori L
- Ribuffo D

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2023

pubdate: 2023 Jan

volume: 11

issue: 1

pages: e4571

abstract: Pseudoangiomatous stromal hyperplasia (PASH) of the breast is a rare and benign medical condition in which the breast tissue is affected by an abnormal myofibroblastic proliferation, which mimics a low-grade sarcoma angiomatous proliferation. PASH usually presents itself either as a palpable mass or as an incidental diagnosis during breast specimens' histological examination. A few cases have been reported in the literature of

a diffuse form of breast PASH syndrome in which the clinical presentation is a bilateral form of gigantomastia without palpable masses. In such cases, the optimal surgical management is still debated due to a significant risk of relapse after breast reduction. Mastectomy seems to be the endpoint of this condition in relapsing cases. Recent studies report a good outcome with a Tamoxifen regimen when surgery cannot be performed, supporting a hormonal component for the etiology of the condition. This study reports on an extremely rare case of bilateral, rapid, and severe PASH in a young patient, presenting as a truly disabling gigantomastia that forced the patient to use a wheelchair due to the excessive breast weights (25 kg the right breast and 21 kg the left). We describe her complicated medical history, her diagnosis, and our course of treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9872972/

Record 560

pmcid: PMC10395857

pmid: 37538218

doi: 10.4103/jwas.jwas_30_23

title: Reduction Mammoplasty for Macromastia: Our Experience Using the Inferior Pedicle with Inverted-T Skin Resection.

authors

- Yiltok SJ
- Akintayo AJ
- Karago CY
- Choji JD
- Sankey B
- Agada E
- Dafong AA
- Ezekiel AS
- Orkar KS

journal: Journal of the West African College of Surgeons

journalAbbr: J West Afr Coll Surg

year: 2023

pubdate: 2023 Jul-Sep

volume: 13

issue: 3

pages: 77-83

abstract: INTRODUCTION: Macromastia is a major reason why women seek for breast reduction especially when there are symptoms. Breast reduction is an uncommon procedure in our environment and this is a report of our experience with inferior pedicle with an inverted-T skin resection. MATERIALS AND METHODS: This is a report of breast reductions using the inferior pedicle with an inverted-T skin resection approach that were carried out between 2004 and 2022. The information that was retrieved from the case notes were age, marital status, parity, last child birth, family history of breast enlargement, presenting features, height, weight, the weight of tissue excised, and complications. The data obtained were then entered into the SPSS version 25 (IBM Corp.) software and analysed. RESULTS: Seventeen women were managed with an age range of 16-50 years and a mean of 31.06 ± 9.66 years. The most common features at presentation were heavy weight around the chest, backache, shoulder pain, no appropriate brassiere with low self-esteem and self-confidence. Ten had skin changes and recurrent rashes over the breast, whereas four complained of grooves with skin changes. The weight of excised breast tissue ranged from 0.2 to 5.5 kg with an average of 2.18 ± 1.28 kg for the right and 2.05 ± 1.00 kg for the left breast. All except one patient had blood transfusion. The common complications were delayed wound healing (47%) followed by partial wound dehiscence (17.6%), and flap necrosis (11.8%). Except

for those with flap necrosis the wounds healed with some having broad scars. CONCLUSIONS: Patients for breast reduction are mainly due to the symptoms and signs with the associated large breast. The inferior pedicle with inverted-T skin resection is a valuable technique with a very good outcome. To improve access to breast reduction, there is the need to enhance awareness through advocacy using women groups and health education.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10395857/

Record 561

pmcid: PMC10162872

pmid: 37153692

doi: 10.1155/2023/6615789

title: Relationship between Body Mass Index and Bone Turnover Markers in Girls with Idiopathic Central Precocious Puberty.

authors

- Zhang J
- Zhou WJ
- Zhang YD
- Liu CJ
- Yu F
- Jiang YM

journal: International journal of clinical practice

journalAbbr: Int J Clin Pract

year: 2023

pubdate: 2023

volume: 2023

issue:

pages: 6615789

abstract: BACKGROUND: This study aimed to determine the effect of body mass index (BMI) on bone turnover markers in girls with idiopathic central precocious puberty (ICPP) according to weight status at diagnosis. METHODS: Two hundred and eleven girls with ICPP were divided according to their weight status at diagnosis into three groups: normal weight, overweight, and obese. The serum levels of total procollagen type 1 N-terminal propeptide (P1NP), N-terminal midfragment of osteocalcin, β -C-terminal telopeptide of type 1 collagen, and some biochemical indicators were measured. Associations between variables were evaluated by multiple regression analysis. RESULTS: Serum P1NP concentrations were significantly different among groups ($p < 0.001$). No other significant differences were noted in N-terminal midfragment of osteocalcin and β -C-terminal telopeptide of type 1 collagen. BMI was associated with estradiol ($r = 0.155$, $p < 0.05$) and inversely associated with P1NP ($r = -0.251$, $p < 0.01$), luteinizing hormone peak ($r = -0.334$, $p < 0.01$), follicle-stimulating hormone peak ($r = -0.215$, $p < 0.01$), and luteinizing hormone/follicle-stimulating hormone peak ($r = -0.284$, $p < 0.01$). Multiple regression analysis of factors associated with BMI showed that it was correlated with P1NP, follicle-stimulating hormone base, and luteinizing hormone peak in the overweight group and the obese group. CONCLUSIONS: Our findings showed that BMI was associated with P1NP, revealing the reduction of bone formation in overweight and obese girls with ICPP. During the diagnosis and treatment of girls with ICPP, attention should be paid to body weight and bone metabolism.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10162872/

Record 562

pmcid: PMC9976717

pmid: 36873230

doi: 10.1259/bjrcr.20220074

title: Repeated recurrence of bilateral gigantomastia after subcutaneous mastectomy caused by tumoral pseudoangiomatous stromal hyperplasia: a case report and review of literature.

authors

- Sornlertlumvanich M
- Rohitopakarn P
- Samphao S
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- Danglaoun S

journal: BJR case reports

journalAbbr: BJR Case Rep

year: 2023

pubdate: 2023 Feb

volume: 9

issue: 1

pages: 20220074

abstract: Pseudoangiomatous stromal hyperplasia (PASH) is an uncommon benign proliferative mesenchymal lesion of the breast with a hormonal-sensitive nature. Various manifestations of PASH, ranging from an incidental microscopic finding in a tissue biopsy to a large palpable mass or bilateral gigantomastia, have been described. For tumoral PASH, surgical excision is indicated for a growing and symptomatic mass with a small chance of recurrence. A recurrence of bilateral gigantomastia after surgical excision or reduction mammoplasty is not common but has been occasionally reported, leading to further mastectomy. Repeated recurrence of bilateral gigantomastia is extremely rare. Herein, we report a case of a 13-year-old girl who presented with the third recurrence of bilateral gigantomastia caused by tumoral PASH, after undergoing bilateral reduction mammoplasty, and later subcutaneous mastectomy. Precocious puberty occurred early in this child at the age of 9 years, which may have been a factor unmasking PASH at this young age. The incomplete removal of the PASH could also have been a recurrence risk in our case as extended masses underneath the pectoralis muscle were later identified on the MRI study. This highlights the advantage of preoperative imaging in cases with a very large tumoral PASH in order to maximize the chance of complete tumor removal.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9976717/

Record 563

pmcid: PMC9882618

pmid: 36712946

doi: 10.2147/DDDT.S393480

title: Research Progress on the Efficacy and Safety of Spironolactone in Reversing Left Ventricular Hypertrophy in Hemodialysis Patients.

authors

- Sun Z
- Chen Z
- Liu R

- Lu G
- Li Z
- Sun Y

journal: Drug design, development and therapy

journalAbbr: Drug Des Devel Ther

year: 2023

pubdate: 2023

volume: 17

issue:

pages: 181-190

abstract: The mineralocorticoid receptor antagonist spironolactone has been shown to improve cardiac function and reverse left ventricular hypertrophy in heart failure patients, but there are no consistent findings on the efficacy and safety in hemodialysis patients. Abnormal aldosterone secretion plays a critical role in the formation of left ventricular hypertrophy. Because of the existence of "aldosterone escape", the routine use of angiotensin-converting enzyme inhibitors/angiotensin receptor blockers does not completely inhibit aldosterone secretion. Low-dose spironolactone (25 mg/d) has been found in small-sample clinical studies to have a significant positive impact with respect to decreasing left ventricular mass index, increasing left ventricular ejection fraction, reversing left ventricular hypertrophy, and improving cardiovascular function while still being safe. More prospective multicenter clinical trials with large sample sizes are needed, however, to provide convincing evidence.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9882618/

Record 564

pmcid: PMC10596285

pmid: 37881167

doi: 10.2147/RMHP.S431228

title: Risk Factors for Granulomatous Mastitis and Establishment and Validation of a Clinical Prediction Model (Nomogram).

authors

- Zeng Y
- Zhang D
- Fu N
- Zhao W
- Huang Q
- Cui J
- Chen Y
- Liu Z
- Zhang X
- Zhang S
- Mansoor KM

journal: Risk management and healthcare policy

journalAbbr: Risk Manag Healthc Policy

year: 2023

pubdate: 2023

volume: 16

issue:

pages: 2209-2222

abstract: BACKGROUND: This study aimed to explore the risk factors and clinical characteristics of granulomatous mastitis (GM) using a case-control study and establish and validate a clinical prediction model (nomogram). METHODS: This retrospective case-control study was conducted in three hospitals in China from June 2017 to December 2021. A total of 1634 GM patients and 186 healthy women during the same period were included and randomly divided into the modeling and validation groups in a 7:3 ratio. To identify the independent risk factors of GM, univariate and multivariate logistic analyses were conducted and used to develop a nomogram. The prediction model was internally and externally validated using the Bootstrap technique and validation cohort. The receiver operating characteristic (ROC) curve and calibration curve were used to evaluate the discrimination and calibration of the prediction model. Decision curve analysis (DCA) and clinical impact curve (CIC) were used to evaluate the clinical significance of the model. RESULTS: The average age of GM patients was 33.14 years (mainly 20-40). The incidence was high within five years from delivery and mainly occurred in the unilateral breast. The majority of the patients exhibited local skin alterations, while some also presented with systemic symptoms. On multivariate logistic analysis, age, high prolactin level, sex hormone intake, breast trauma, nipple discharge or invagination, and depression were independent risk factors for GM. The mean area under the curve (AUC) in the modeling and validation groups were 0.899 and 0.889. The internal and external validation demonstrated the model's predictive ability and clinical value. CONCLUSION: Lactation-related factors are the main risk factors of GM, leading to milk stasis or increased ductal secretion. Meanwhile, hormone disorders could affect the secretion and expansion of mammary ducts. All these factors can obstruct or injure the duct, inducing inflammatory reactions and immune responses. Additionally, blunt trauma, depressed mood, and diet preference can accelerate the process. The nomogram can effectively predict the risk of GM.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10596285/

Record 565

pmcid: PMC9985532

pmid: 36749528

doi: 10.1007/s40268-023-00413-x

title: Risk of Adverse Events Associated with Domperidone and Metoclopramide in Gastroparesis: Systematic Review and Meta-analysis.

authors

- Junqueira DR
- Bennett D
- Huh SY
- Fahrbach K
- Neupane B
- Betts M

journal: Drugs in R&D

journalAbbr: Drugs R D

year: 2023

pubdate: 2023 Mar

volume: 23

issue: 1

pages: 1-20

abstract: BACKGROUND: Dopamine antagonists are the main pharmacological options to treat gastroparesis. The aim of this study was to conduct a systematic literature review (SLR) to evaluate the profile of adverse events (AEs) of dopamine antagonists used in the treatment of children and adults with gastroparesis. METHODS: We searched EMBASE and MEDLINE up to March 25, 2021, for relevant clinical trials and observational studies. We conducted a proportional meta-analysis to estimate the pooled occurrence of AEs (%), with 95% confidence interval (CI), from arm-level data across studies and the comparative occurrence of AEs from placebo-controlled clinical trials (odds ratio [OR] with 95% CI). RESULTS: We identified 28 studies assessing AEs experienced by patients treated for gastroparesis with domperidone and metoclopramide; 22 studies contributed data to the meta-analyses. Cardiovascular, neurological, and endocrine AEs were commonly observed, with point incidences varying from 1 to > 50%. Clinically important AEs, such as QTc prolongation, occurred in 5% of patients treated with domperidone (95% CI: 3.32-8.62). Restlessness, an extrapyramidal AE, occurred in 15% of patients (95% CI: 7.48-26.61) treated with metoclopramide, with a 7-fold increase compared with patients receiving placebo (OR: 7.72; 95% CI: 1.27-47.05). Variation in terminology to describe extrapyramidal events precluded further pooled analyses. Additional meta-analyses were not feasible due to discrepancies in the assessment and reporting of the AEs. CONCLUSIONS: The evidence confirms concerns of cardiovascular, extrapyramidal, and endocrine AEs in patients with gastroparesis treated with domperidone and metoclopramide. Imprecise AE reporting limits firm interpretation and conclusions. REGISTRATION: PROSPERO international prospective register of systematic reviews (registration number: CRD42021248888).

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9985532/

Record 566

pmcid: PMC10072151

pmid: 37025099

doi: 10.2147/DHPS.S383130

title: Safety Aspects and Rational Use of Testosterone Undecanoate in the Treatment of Testosterone Deficiency: Clinical Insights.

authors

- Campbell K
- Muthigi A
- Ghomeshi A
- Schuppe K
- Sandler MD
- Ramasamy R

journal: Drug, healthcare and patient safety

journalAbbr: Drug Healthc Patient Saf

year: 2023

pubdate: 2023

volume: 15

issue:

pages: 73-84

abstract: Testosterone deficiency is diagnosed by a serum total testosterone level below 300 ng/dL in combination with symptoms such as decreased energy and libido. These symptoms can be ameliorated by restoring serum testosterone to the physiologic range with testosterone therapy (TT). There are numerous forms of testosterone therapy, such as injectable, transdermal, nasal, and subcutaneous applications. There are also multiple formulations of injection, such as testosterone cypionate, testosterone enanthate, and testosterone undecanoate. Testosterone undecanoate (TU) is a long-acting ester formulation of testosterone that can be provided in an injectable or oral form. Oral testosterone undecanoate is marketed as Andriol, Jatenzo, Tlando, and Kyzatrex. Oral TU provides a convenient option for many patients, which may increase compliance with TT.

Injectable testosterone undecanoate is marketed as Aveed and Nebido. Injectable TT remains the most cost-effective therapeutic option and is appropriate for most patients as an initial therapy. This review describes the pharmacokinetics of these testosterone undecanoate products and provides a guide for prescribers using these medications. While many forms of testosterone are appropriate for TT, a patient-centered discussion focused on goals of care should best guide physician prescription of these medications.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10072151/

Record 567

pmcid: PMC10257295

pmid: 37296473

doi: 10.1186/s13058-023-01667-8

title: Studying the association between longitudinal mammographic density measurements and breast cancer risk: a joint modelling approach.

authors

- Illipse M
- Czene K
- Hall P
- Humphreys K

journal: Breast cancer research : BCR

journalAbbr: Breast Cancer Res

year: 2023

pubdate: 2023 Jun 9

volume: 25

issue: 1

pages: 64

abstract: BACKGROUND: Researchers have suggested that longitudinal trajectories of mammographic breast density (MD) can be used to understand changes in breast cancer (BC) risk over a woman's lifetime. Some have suggested, based on biological arguments, that the cumulative trajectory of MD encapsulates the risk of BC across time. Others have tried to connect changes in MD to the risk of BC. METHODS: To summarize the MD-BC association, we jointly model longitudinal trajectories of MD and time to diagnosis using data from a large ([Formula: see text]) mammography cohort of Swedish women aged 40-80 years. Five hundred eighteen women were diagnosed with BC during follow-up. We fitted three joint models (JMs) with different association structures; Cumulative, current value and slope, and current value association structures. RESULTS: All models showed evidence of an association between MD trajectory and BC risk ([Formula: see text] for current value of MD, [Formula: see text] and [Formula: see text] for current value and slope of MD respectively, and [Formula: see text] for cumulative value of MD). Models with cumulative association structure and with current value and slope association structure had better goodness of fit than a model based only on current value. The JM with current value and slope structure suggested that a decrease in MD may be associated with an increased (instantaneous) BC risk. It is possible that this is because of increased screening sensitivity rather than being related to biology. CONCLUSION: We argue that a JM with a cumulative association structure may be the most appropriate/biologically relevant model in this context.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10257295/

Record 568

pmcid: PMC10448552

pmid: 34965699

doi: 10.4274/jcrpe.galenos.2021.2021.0256

title: Syndrome of Congenital Insulin Resistance Caused by a Novel INSR Gene Mutation.

authors

- Rojek A
- Wikiera B
- Noczynska A
- Niedziela M

journal: Journal of clinical research in pediatric endocrinology

journalAbbr: J Clin Res Pediatr Endocrinol

year: 2023

pubdate: 2023 Aug 23

volume: 15

issue: 3

pages: 312-317

abstract: Mutations in the INSR gene result in rare inherited syndromes causing insulin resistance, such as leprechaunism (Donohue syndrome), Rabson-Mendenhall syndrome and insulin resistance type A. Leprechaunism is an autosomal recessive disorder associated with extreme insulin resistance that leads to hyperinsulinemia, impaired glucose homeostasis, fasting hypoglycemia and postprandial hyperglycemia. Impaired insulin action causes prenatal and postnatal growth retardation. Lipoatrophy, dysmorphic facies, hypertrichosis, macrogenitosomia and hypertrophy of internal organs are also present. A male infant with congenital insulin resistance was born at term after a normal pregnancy with a weight of 1905 g (<3 c), a length of 48 cm (<3 c), and an Apgar score of 10. Intrauterine growth retardation, transient hypoglycemia, pneumonia, urinary tract infection and heart defects [patent foramen ovale (PFO); patent ductus arteriosus (PDA)] were diagnosed after birth. At 5 weeks of age, he was admitted to the regional hospital with severe fever, diarrhea and dehydration. Hyperglycemia was observed (672 mg/dL), and insulin was administered. He was referred to a hospital at 7 weeks of age for suspected neonatal diabetes and hypertrophic cardiomyopathy. The physical examination revealed a loud systolic heart murmur, tachycardia, tachypnea, dysmorphic facies, hypertrichosis, acanthosis nigricans, hypotonia, swollen nipples and enlarged testicles. Glycemic fluctuations (50-250 mg/dL) were observed. The serum insulin concentration was high (maximum 1200 IU/mL) at normoglycemia. Ultrasound of the heart confirmed progressive hypertrophic cardiomyopathy. Leprechaunism was confirmed by genetic analysis of INSR , in which a novel c.320C>G; p. Thr107Arg homozygous missense mutation was found in exon 2.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10448552/

Record 569

pmcid: PMC10221400

pmid: 37240986

doi: 10.3390/jpm13050816

title: Textured vs. Smooth Breast Implants Using the Jones Criteria-What Is the Currently Available Evidence for BIA-ALCL?: A Systematic Review.

authors

- Hecker A
- Giese B
- Pignet AL
- Schellnegger M
- Kamolz LP

- Lumenta DB

  journal: Journal of personalized medicine

  journalAbbr: J Pers Med

  year: 2023

  pubdate: 2023 May 11

  volume: 13

  issue: 5

  pages:

  abstract: UNLABELLED: Breast-Implant-Associated Anaplastic Large-Cell Lymphoma (BIA-ALCL) is a rare low-incidence type of T-cell non-Hodgkin lymphoma, arising in the capsule around breast implants, and predominantly associated with the use of macro-textured breast implants. The purpose of this study was to use an evidence-based approach to systematically identify clinical studies comparing smooth and textured breast implants in women with regard to the risk of developing BIA-ALCL. METHODS: A literature search in PubMed in April 2023 and the article reference list of the French National Agency of Medicine and Health Products decision from 2019 were screened for applicable studies. Only clinical studies where the Jones surface classification could be applied (required information: breast implant manufacturer) for comparison of smooth and textured breast implants were considered. RESULTS: From a total of 224 studies, no articles were included due to the lack of fit to the strict inclusion criteria. CONCLUSIONS: Based on the scanned and included literature, implant surface types in relation to the incidence of BIA-ALCL were not evaluated in clinical studies and data from evidence-based clinical sources plays a minor to no role in this context. An international database that combines breast implant-related data from (national, opt-out) medical device registries is, therefore, the best available option to obtain relevant long-term breast implant surveillance data on BIA-ALCL.

  url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10221400/

Record 570

pmcid: PMC10307648

pmid: 36876744

doi: 10.5534/wjmh.221027

title: The British Society for Sexual Medicine Guidelines on Male Adult Testosterone Deficiency, with Statements for Practice.

authors

- Hackett G
- Kirby M
- Rees RW
- Jones TH
- Muneer A
- Livingston M
- Ossei-Gerning N
- David J
- Foster J
- Kalra PA
- Ramachandran S

  journal: The world journal of men's health

  journalAbbr: World J Mens Health

  year: 2023

pubdate: 2023 Jul

volume: 41

issue: 3

pages: 508-537

abstract: Testosterone deficiency (TD) is an increasingly common problem with significant health implications, but its diagnosis and management can be challenging. A multi-disciplinary panel from BSSM reviewed the available literature on TD and provide evidence-based statements for clinical practice. Evidence was derived from Medline, EMBASE and Cochrane searches on hypogonadism, testosterone therapy (T Therapy) and cardiovascular safety from May 2017 to September 2022. This revealed 1,714 articles, including 52 clinical trials and 32 placebo-controlled randomised controlled trials. A total of twenty-five statements are provided, relating to five key areas: screening, diagnosis, initiating T Therapy, benefits and risks of T Therapy, and follow-up. Seven statements are supported by level 1 evidence, eight by level 2, five by level 3, and five by level 4. Recent studies have demonstrated that low levels of testosterone in men are associated with increased risk of incident type 2 diabetes mellitus, worse outcomes in chronic kidney disease and COVID 19 infection with increased all-cause mortality, along with significant quality of life implications. These guidelines should help practitioners to effectively diagnose and manage primary and age-related TD.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10307648/

Record 571

pmcid: PMC10258612

pmid: 37313241

doi: 10.17925/EE.2023.19.1.98

title: The Clinical Septet of Van Wyk-Grumbach Syndrome: A Case Series from a Tertiary Care Centre in Kalyana Karnataka, India.

authors

- Waddankeri S
- Waddankeri M
- Waddankeri S
- Arora K

journal: TouchREVIEWS in endocrinology

journalAbbr: touchREV Endocrinol

year: 2023

pubdate: 2023 May

volume: 19

issue: 1

pages: 98-102

abstract: Van Wyk-Grumbach syndrome is a rare, female juvenile hypothyroidism disorder that is characterized by precocious puberty with clinical, radiological and hormonal pathologies. We present a case series of three patients with this unusual condition who were evaluated and followed up over a 3-year period between January 2017 and June 2020. All three patients presented with short stature (<3rd centile), low weight (<3rd centile), absence of goitre, no axillary or pubic hair, delayed bone age by more than 2 years, elevated thyroid-stimulating hormone with low T3 and T4 (primary hypothyroidism), and raised follicle-stimulating hormone with pre-pubertal levels of luteinizing hormone. Abdominal ultrasonography showed bilateral multi-cystic ovaries in two patients and a right-sided bulky ovary in the third patient. One of the patients also had a pituitary 'macroadenoma'. All the patients were successfully managed with levothyroxine. We discuss the pathophysiological mechanisms with a brief literature review.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10258612/

Record 572

pmcid: PMC10796121

pmid: 38239913

doi: 10.1080/23802359.2023.2275827

title: The complete plastome of Campsis radicans (L.) Bureau 1864 and its phylogenetic analysis.

authors

- Li H
- Wang L
- Ma C

journal: Mitochondrial DNA. Part B, Resources

journalAbbr: Mitochondrial DNA B Resour

year: 2023

pubdate: 2023

volume: 8

issue: 11

pages: 1200-1204

abstract: Campsis radicans (L.) Bureau 1864, a species of Bignoniaceae, has a widespread paleotropical distribution and is utilized for horticultural and traditional Chinese medicinal purposes. Despite the plant's significance, its genetic diversity must be better understood. In this study, we have successfully assembled and characterized the complete plastome of C. radicans , marking a significant advancement toward comprehending its genetic composition. The plastome is 153,630 bp long and harbors 130 genes, including 86 protein-coding genes, 36 tRNA genes, and eight rRNA genes. Our phylogenomic analysis of the representative species of Bignoniaceae indicated that C. radicans formed a monophyletic sister clade of Campsis with C. grandiflora . These findings are crucial for conserving and utilizing this important plant species. They also highlight the potential for future research into the evolution and preservation of C . radicans , which could be advantageous in pharmaceutical applications.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10796121/

Record 573

pmcid: PMC10354431

pmid: 37476374

doi: 10.3389/fonc.2023.1174173

title: The evaluation of the delayed swollen breast in patients with a history of breast implants.

authors

- Keane GC
- Keane AM
- Diederich R
- Kennard K
- Duncavage EJ
- Myckatyn TM

journal: Frontiers in oncology

journalAbbr: Front Oncol

year: 2023

pubdate: 2023

volume: 13

issue:

pages: 1174173

abstract: Breast implants, whether placed for reconstructive or cosmetic purposes, are rarely lifetime devices. Rupture, resulting from compromised implant shell integrity, and capsular contracture caused by constriction of the specialized scar tissue that normally forms around breast implants, have long been recognized, and remain the leading causes of implant failure. It is apparent, however, that women with breast implants may also experience delayed breast swelling due to a range of etiologic factors. While a majority of delayed seromas associated with breast implants have a benign etiology, this presentation cannot be ignored without an adequate workup as malignancies such as breast implant associated anaplastic large cell lymphoma (BIA-ALCL), breast implant associated diffuse large B-cell lymphoma (BIA-DLBCL), and breast implant associated squamous cell carcinoma (BIA-SCC) can have a similar clinical presentation. Since these malignancies occur with sufficient frequency, and with sometimes lethal consequences, their existence must be recognized, and an appropriate diagnostic approach implemented. A multidisciplinary team that involves a plastic surgeon, radiologist, pathologist, and, as required, surgical and medical oncologists can expedite judicious care. Herein we review and further characterize conditions that can lead to delayed swelling around breast implants.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10354431/

Record 574

pmcid: PMC9614759

pmid: 36306074

doi: 10.1007/s40592-022-00164-6

title: The final frontier: what is distinctive about the bioethics of space missions? The cases of human enhancement and human reproduction.

authors

- Szocik K
- Reiss MJ

journal: Monash bioethics review

journalAbbr: Monash Bioeth Rev

year: 2023

pubdate: 2023 Dec

volume: 41

issue: 2

pages: 87-102

abstract: We examine the bioethical issues that arise from long-duration space missions, asking what there is that is distinctive about such issues. We pay particular attention to the possibility that such space missions, certainly if they lead to self-sustaining space settlements, may require human enhancement, and examine the significance of reproduction in space for bioethics. We conclude that while space bioethics raises important issues to do with human survival and reproduction in very hazardous environments, it raises no issues that are distinct from those in terrestrial bioethics. Rather, space bioethics raises extreme versions of bioethical issues that are already found in the military, when working in extreme environments (such as Antarctica), or when living in circumstances (such as in prison) where one's autonomy is severely curtailed.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9614759/

Record 575

pmcid: PMC10328791

pmid: 37425607

doi: 10.7759/cureus.40104

title: Total Vaginectomy in a True Hermaphrodite: A Case Report.

authors

- Das S
- Swain UR

journal: Cureus

journalAbbr: Cureus

year: 2023

pubdate: 2023 Jun

volume: 15

issue: 6

pages: e40104

abstract: A case is reported herein of a true hermaphrodite (TH) with an ovotestis, a uterus, a vagina, and an underdeveloped phallus. The patient was raised by his parents as a male, based on the presence of a phallus with ambiguous genitalia. He started experiencing breast enlargement at the age of 14 and menarche by the age of 17. He was reviewed using ultrasound, magnetic resonance imaging of the abdomen, and karyotyping, and the reports showed evidence of Mullerian structures and 46 XX karyotyping. Based on the preferences of the patient and his parents and their psychological outlook toward the male gender, a total mastectomy, hysterectomy, bilateral gonadectomy, and total vaginectomy were performed. This was followed by reconstruction of the male genitalia and supplemented with male hormone replacement therapy. Accordingly, a TH was assigned a male gender.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10328791/

Record 576

pmcid: PMC10404866

pmid: 37554126

doi: 10.1016/j.eclinm.2023.102113

title: Toxicity profile of antibody-drug conjugates in breast cancer: practical considerations.

authors

- D'Arienzo A
- Verrazzo A
- Pagliuca M
- Napolitano F
- Parola S
- Viggiani M
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- Del Mastro L
- Arpino G

- De Laurentiis M
- Montemurro F

journal: EClinicalMedicine

journalAbbr: EClinicalMedicine

year: 2023

pubdate: 2023 Aug

volume: 62

issue:

pages: 102113

abstract: Antibody-drug conjugates (ADCs) represent a novel and evolving class of antineoplastic agents, constituted by monoclonal antibody linked to biologically active drugs, delivering cytotoxic compounds at the tumor site, reducing the likelihood of systemic exposure and toxicity. They are generally well tolerated, nevertheless some predictable adverse reactions need careful monitoring and timely approach. These include neutropenia, nausea and vomiting, alopecia, diarrhea, left ventricular dysfunction, ILD/pneumonitis. The mechanisms leading to drug-associated toxicities are summarized, and prophylaxis protocols and appropriate management strategies are proposed, based on current literature. This review aims to collect the most updated evidence on toxicities potentially occurring during breast cancer treatment with approved or under clinical investigation (advanced stage) ADCs. A focus is dedicated to monitoring protocols and clinical management, aimed at preventing and/or promptly address relevant problems, in order to avoid premature discontinuation or improper dose reduction.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10404866/

Record 577

pmcid: PMC10090427

pmid: 37064139

doi: 10.3389/fonc.2023.1157545

title: Trans-axillary single port insufflation technique-assisted endoscopic surgery for breast diseases: Clinic experience, cosmetic outcome and oncologic result.

authors

- Wang X
- Wan X
- Li L
- Liu X
- Meng R
- Sun X
- Xiao C

journal: Frontiers in oncology

journalAbbr: Front Oncol

year: 2023

pubdate: 2023

volume: 13

issue:

pages: 1157545

abstract: PURPOSE: With an increasing demand for postoperative cosmetic effects in breast diseases, the single port by trans-axillary incision and air-inflation system, which provided better space and spared the assistant the effort of retraction, is widely used in clinic surgical treatment for multiple breast diseases. METHODS: According to inclusion and exclusion criteria, patients who underwent trans-axillary single-incision surgery at Tianjin Medical University Cancer Hospital between December 2020 and July 2022 were included in the study. We collected and analyzed data on age, fertility history, ultrasound grade, clinical stage, pathological results, oncological prognosis, patient-centered cosmetic outcome, etc. RESULTS: A total of 115 cases were included, of which 33 patients with benign disease underwent mass resection, 68 patients with malignant tumors underwent mastectomy. 10 patients had a special type of breast lesion. A mastectomy was performed in 4 patients with male mammary gland development. Of the 115 cases, the maximum mass diameter was 3.00 ± 1.644 (0.6-8.5) cm. Blood loss during surgery was 85.77 ± 50.342 (10-200) ml. The surgery took 131.84 ± 59.332 (30-280) minutes to complete. The patient spent a total of 5.05 ± 2.305 (2-18) days in the hospital. And the length of surgical incision in all patients was 3.83 ± 0.884 (3-8) cm. All patients were very satisfied with the appearance of their breasts after dressing. 94.78% of patients were satisfied with the position of the incision. CONCLUSION: Through this study, we believe that in benign breast diseases and malignant breast tumors, trans-axillary single port insufflation technique-assisted endoscopic surgery has oncological safety and an aesthetic effect for most people with breast diseases.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10090427/

Record 578

pmcid: PMC9991427

pmid: 36895310

doi: 10.1089/trgh.2021.0023

title: Transgender and Alagille Syndrome: A Rare Case of a Trans Woman with Alagille Syndrome.

authors

- Shefler H
- Berl A
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journal: Transgender health

journalAbbr: Transgend Health

year: 2023

pubdate: 2023 Feb

volume: 8

issue: 1

pages: 108-111

abstract: Alagille syndrome is a rare autosomal dominant disorder with variable expression. Liver damage, especially cholestatic, is the most common feature of the syndrome. Transgender patients may suffer from a great distress due to the discrepancy between assigned sex at birth and unaffirmed gender identity. Gender affirmation treatment options for these patients include hormone therapy (HT) to induce secondary sexual characteristics and various surgical procedures. Estrogen-based hormonal treatments have been linked to an increased risk of liver enzyme elevation and disruption of bilirubin metabolism, especially in those with a genetic susceptibility. The case presented here is the first described Alagille syndrome transgender patient to undergo gender affirmation treatment, including (HT) and vulvo-vaginoplasty surgery.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9991427/

Record 579

pmcid: PMC10210567

pmid: 37092615

doi: 10.23750/abm.v94i2.14324

title: Unilateral breast enlargement in males during adolescence (10-19 years): Review of current literature and personal experience.

authors

- De Sanctis V
- Soliman AT
- Tzoulis P
- Daar S
- Di Maio S
- Kattamis C

journal: Acta bio-medica : Atenei Parmensis

journalAbbr: Acta Biomed

year: 2023

pubdate: 2023 Apr 24

volume: 94

issue: 2

pages: e2023004

abstract: Idiopathic unilateral breast enlargement (UBE) in males is a, commonly overlooked, diagnosis of exclusion that requires careful history, meticulous physical examination, and pertinent laboratory studies to exclude the possible pathologic causes. The aims of the present update are to review the current literature on UBE in subjects during adolescent age (10-19 years) in 18 cases, and to report the personal experience in 13 adolescents referred to our unit during the last four decades. In total, our survey and personal experience include 31 UBE cases, 10 of whom (32.2 %) being idiopathic or familial gynecomastia (GM). In 3/31 (9.6%) UBE was due to breast sarcoma/ carcinoma; one patient (11-years old) had a 5-year history of painless lump in the right breast, which increased gradually in size followed by bloody nipple discharge. In the personal cases of 13 adolescents, a moderate to marked UBE was secondary to: treatment with androgens (2 β-thalassemic patients with hypogonadism), high estrogen/androgen ratio in 2 Klinefelter syndrome patients, peripheral aromatization of androgens in 1 patient with non-classical 21-hydroxylase deficiency (NC-21-OH-D). One patient had subareolar hematoma due to injury. In 2 patients (15,3%) marked UBE was due to cystic lymphangioma (histologically proved). Furthermore, 5 patients were characterized as idiopathic UBE In clinical practice, the persistence of UBE for long period before diagnosis necessitates attention and further evaluation. Underlying causes should be treated, when possible, while surgery can be offered to patients with persistent or atypical signs and/or symptoms of UBE. For the optimal management of this condition, better collaboration between primary care physician and specialists is mandatory.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10210567/

Record 580

pmcid: PMC10371879

pmid: 37071372

doi: 10.1007/s40618-023-02077-5

title: Update on bioethical, medical and fertility issues in gender incongruence during transition age.

authors

- Conflitti AC
- Spaziani M

- Pallotti F
- Tarsitano MG
- Di Nisio A
- Paoli D
- Lombardo F

journal: Journal of endocrinological investigation

journalAbbr: J Endocrinol Invest

year: 2023

pubdate: 2023 Sep

volume: 46

issue: 9

pages: 1725-1736

abstract: PURPOSE: Many issues still remain unresolved in the management of pubertal patients with gender incongruence (GI). The aim of this review is to discuss the main aspects of the treatment of these patients to provide a practical approach for clinicians. METHODS: A comprehensive literature search within PubMed was performed to provide updates of available evidence regarding the impact on bioethical, medical and fertility issues in gender incongruence during transition age. RESULTS: Gender Affirming Hormone Treatment (GAHT) and Gender Affirming Surgery (GAS) can induce unsatisfaction with change, future regrets, and the risk of infertility. This raises ethical issues especially in the management of pubertal patients that remain unresolved. Therapy with GnRH analogues (GnRHa) is intended to delay puberty, so as to give the adolescent a longer period of time to decide whether to continue with the treatments. At the level of physical changes, this therapy may have an effect on bone mineralization and body composition; however, long-term longitudinal data are not yet available. An important feature related to the use of GnRHa is the risk of fertility. Gamete cryopreservation is the most established method of fertility preservation (FP) and should be counselled to transgender adolescents. However, these patients are not always interested in having biological children. CONCLUSION: Based on the current evidence, there is a need to conduct further research to clarify certain issues and to standardize clinical practice and improve counselling in transgender adolescent decision making and avoid regrets in the future.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10371879/

Record 581

pmcid: PMC10902889

pmid: 37792608

doi: 10.1093/asj/sjad323

title: Update on Oblique Flankplasty: Easily Executed, Long-Lasting, Integral Component of Total Body Lift Surgery.

authors

- Hurwitz DJ
- Davila AA

journal: Aesthetic surgery journal

journalAbbr: Aesthet Surg J

year: 2023

pubdate: 2023 Dec 14

volume: 44

issue: 1

pages: NP77-NP86

abstract: BACKGROUND: Twenty years ago, coordinated aesthetic surgery for laxity and lipodystrophy after massive weight loss (MWL), so-called total body lift surgery (TBL), encompassed circumferential hip hugging transverse lower body lift (LBL) with possible buttock auto-augmentation, and a transverse bra line upper body lift (UBL) with breast reshaping. Brachioplasty and vertical thighplasty were often included. Disappointing aesthetics of the posterior torso led to innovation with J-torsoplasty and oblique flankplasty. OBJECTIVES: The goal of this study was to demonstrate in a large clinical series and in a range of case presentations from 2 plastic surgeons that oblique flankplasty with lipoabdominoplasty (OFLA) optimally narrows the waist, suspends lateral buttocks and thighs, and integrates with J-torsoplasty and vertical thighplasty to tighten skin and aesthetically contour the torso and thighs with an acceptable rate of complications. METHODS: Retrospective chart review of 151 consecutive flankplasties between June 2010 and April 2023, including sex, age, BMI, associated operations, complications, and revisions was performed. Five case presentations were accompanied by limited photographs and a marking video. RESULTS: Across a broad clinical spectrum, malleable oblique flankplasty resected bulging flanks and, facilitated by neighboring liposuction and/or J-torsoplasty, consistently pulled in lax skin and anchored through cadaver-proven dense dermal adherences lax tissues to create a long-lasting skintight shapely torso and upper thighs, with only 3.3% problematic wounds. Five diverse cases showed broad applicability. CONCLUSIONS: OFLA, often with J-torsoplasty and neighboring liposuction, aesthetically recontours torso skin laxity in a variety of presentations with a low rate of complications in a high-risk population.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10902889/

Record 582

pmcid: PMC10669987

pmid: 38002921

doi: 10.3390/children10111830

title: Use of the Subcutaneous Triptorelin Stimulation Test for Diagnosis of Central Precocious Puberty.

authors

- Ahn J
- Lee Y
- Gim S
- Jeong H

journal: Children (Basel, Switzerland)

journalAbbr: Children (Basel)

year: 2023

pubdate: 2023 Nov 20

volume: 10

issue: 11

pages:

abstract: BACKGROUND: The gold standard gonadotropin-releasing hormone (GnRH) stimulation test uses the response to intravenously injected gonadorelin to diagnose central precocious puberty (CPP). However, gonadorelin is not always readily available. OBJECTIVE: This study investigated the diagnostic efficacy of the subcutaneous triptorelin test and the optimal blood sampling time for diagnosis of CPP. METHODS: This study retrospectively examined the medical records of 220 girls who had undergone either the triptorelin or gonadorelin test and compared their clinical characteristics. We retrospectively compared clinical parameters between girls diagnosed with CPP (n = 111) and idiopathic premature thelarche (IPT) (n = 109) using three different diagnostic methods: the gonadorelin, triptorelin 120 min, and triptorelin 180 min tests. The diagnostic ability of the stimulated luteinizing hormone (LH) concentration in the triptorelin test for CPP was evaluated using receiver operating characteristic (ROC) analysis. RESULTS: The CPP group exhibited higher basal and peak gonadotropin levels, more advanced bone age, and a lower body mass index standard deviation score than the IPT group. In the gonadorelin test group, all girls with CPP exhibited a peak LH response 30-60 min

after intravenous gonadorelin injection. In the triptorelin test group, most girls with CPP exhibited a peak LH response 60-180 min after subcutaneous triptorelin injection (n = 68). On the ROC curve, a peak LH concentration of ≥ 4.52 IU/L at 120 min had the highest CPP diagnostic accuracy, with sensitivity and specificity of 100% and 95.83%, respectively.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10669987/

Record 583

pmcid: PMC10326455

pmid: 37425593

doi: 10.7759/cureus.40067

title: Virginal Breast Hypertrophy: A Case Report.

authors

- Soliman SA
- Algatheradi MA
- Aljahwashi TA
- Alhussan TH
- Alqahtani RS
- Ali NI

journal: Cureus

journalAbbr: Cureus

year: 2023

pubdate: 2023 Jun

volume: 15

issue: 6

pages: e40067

abstract: Virginal breast hypertrophy, also known as juvenile macromastia or juvenile gigantomastia, is an uncommon condition characterized by the rapid and excessive growth of breasts in prepubertal or peripubertal girls in the absence of any hormonal or physiological causes. While virginal breast hypertrophy is a rare benign disorder that occurs independent of hormonal stimulation, it can cause a diagnostic challenge to physicians and requires a multidisciplinary team to get it right. It also results in detrimental effects, both physical and psychological, for young girls. We present a case of virginal breast hypertrophy in an 11-year-old Saudi girl, which was successfully managed. This report will contribute to knowledge sharing with healthcare professionals in Saudi Arabia about this rare case. It can also pave roads for further research to understand the underlying mechanisms and to standardize treatment modalities.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10326455/

Record 584

pmcid: PMC10581915

pmid: 37337055

doi: 10.1007/s00266-023-03420-0

title: Watermelons, Syringes, and Regulation: Instagram Marketing by Cosmetic Clinics in the UK and the Netherlands.

authors

- Hermans AM

journal: Aesthetic plastic surgery

journalAbbr: Aesthetic Plast Surg

year: 2023

pubdate: 2023 Oct

volume: 47

issue: 5

pages: 2179-2189

abstract: BACKGROUND: Cosmetic clinics use social media to inform and market to prospective patients. Evidence from social media posts by the UK and Dutch cosmetic clinics illustrates the precarious balance between professionalism and commercialism, and raises important questions for the regulation of these marketing communication efforts. METHODS: A random selection of Instagram posts by Dutch and the UK clinics which offer (non-) invasive cosmetic procedures were selected for a qualitative content analysis. The corpus of data comprised 395 posts by six Dutch and four UK clinics, published between January 2018 and July 2019. The method of analysis was inspired by previous qualitative studies into the marketing of cosmetic procedures and can be described as a (discursive) thematic analysis. RESULTS AND CONCLUSIONS: The tension between the medical-professional and commercial nature of cosmetic procedures was evident in the Instagram posts by Dutch and UK clinics. Despite calls for 'ethical' marketing on social media, this study illustrates that marketing materials are not always in line with current advertising guidelines. Whereas advertising standards authorities have warned against the trivialization of cosmetic procedures, posts by clinics backgrounded the medical nature of procedures in favor of more commercial advertising appeals. Furthermore, the posts demonstrated little diversity in terms of models' gender, ethnicity and body type, which reinforces narrow contemporary beauty ideals. LEVEL OF EVIDENCE V: This journal requires that authors assign a level of evidence to each article. For a full description of these Evidence-Based Medicine ratings, please refer to the Table of Contents or the online Instructions to Authors www.springer.com/00266 .

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10581915/

Record 585

pmcid: PMC11090235

pmid: 38746603

doi: 10.20471/acc.2023.62.s1.17

title: WHAT IS THE RELATIONSHIP BETWEEN A GYNECOLOGIST/OBSTETRICIAN AND THE AIRWAY?

authors

- Habek D
- Miletić AI
- Medić F

journal: Acta clinica Croatica

journalAbbr: Acta Clin Croat

year: 2023

pubdate: 2023 Apr

volume: 62

issue: Suppl1

pages: 132-136

abstract: Physiological changes in pregnancy as part of biohumoral and morphological changes (hyperemia, edema, hypersecretion) influence the possible problems in obstetric anesthesia. These changes by themselves, and particularly aggravated by acute or chronic gestational or non-gestational comorbidity, increase the risk of

aspiration of gastric contents, failed intubation, esophageal intubation, inadequate ventilation, and respiratory failure. The types of premedication, anesthesia and techniques of anesthesia are evident from medical historiography. Almost obligatory promethazine and atropine was given intravenously either in the delivery room or on the operating table immediately before the induction of anesthesia in a dose of 0.5 mg in partuients of average body weight. Atropine has been a favorite premedicant for decades, given its pharmacological properties, especially its antisialogenic effect and absence of a depressant effect on the fetoplacental unit, but today it is rarely used. Nasal decongestants before surgery are not recommended but in cases of severe rhinitis, atropine, promethazine, or topical decongestants may be used.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11090235/

Record 586

pmcid: PMC10400845

pmid: 37524022

doi: 10.1016/j.ijscr.2023.108532

title: Wisepatern mastopexy for natural breast ptosis symmetrization after giant lipoma excision: Case report.

authors

- Yarso KY
- Bellynda M
- Ismail EA
- Kamil MR

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2023

pubdate: 2023 Aug

volume: 109

issue:

pages: 108532

abstract: INTRODUCTION AND IMPORTANCE: Giant breast lipoma is an uncommon benign tumor that develops in the breast parenchyma. Wisepatern technique involves lifting the skin in both vertical and horizontal directions to raise and reshape the breasts into a less ptotic shape. CASE PRESENTATION: A 40-year-old woman came to the Surgical Oncology Polyclinic with the breast size is asymmetric, the right is 4× larger than the left. The patient's request for a tumor removal procedure with a symmetrical approach on the right breast alone, we opted for a Wisepatern surgical technique. Residual skin tissue was discovered and de-epithelialization was performed to remove it, but it was not discarded and instead inserted into the breast cavity to create the effect of a mass filling empty areas left after tumor removal. CLINICAL DISCUSSION: The Wisepatern technique is highly preferred due to its versatility, ease of execution, and ability to achieve consistent outcomes in mastopexy and breast reductions. It effectively addresses excess skin, avoids large dog-ears and longer scars, and allows for a more natural-looking appearance by utilizing a shortened vertical scar and partial subpectoral pocket for implant positioning. CONCLUSION: Mastopexy is a surgical procedure that can effectively treat giant breast lipoma by removing the lipoma and reshaping the breast tissue. However, it is important for patients to be fully informed about the risks and benefits of the procedure and to undergo appropriate follow-up to ensure a successful outcome.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10400845/

Record 587

pmcid: PMC9596528

pmid: 36282442

doi: 10.1007/s40656-022-00533-8

title: "Waking up" the sleeping metaphor of normality in connection to intersex or DSD: a scoping review of medical literature.

authors

- De Clercq E
- Starke G
- Rost M

journal: History and philosophy of the life sciences

journalAbbr: Hist Philos Life Sci

year: 2022

pubdate: 2022 Oct 25

volume: 44

issue: 4

pages: 50

abstract: The aim of the study is to encourage a critical debate on the use of normality in the medical literature on DSD or intersex. For this purpose, a scoping review was conducted to identify and map the various ways in which "normal" is used in the medical literature on DSD between 2016 and 2020. We identified 75 studies, many of which were case studies highlighting rare cases of DSD, others, mainly retrospective observational studies, focused on improving diagnosis or treatment. The most common use of the adjective normal was in association with phenotypic sex. Overall, appearance was the most commonly cited criteria to evaluate the normality of sex organs. More than 1/3 of the studies included also medical photographs of sex organs. This persistent use of normality in reference to phenotypic sex is worrisome given the long-term medicalization of intersex bodies in the name of a "normal" appearance or leading a "normal" life. Healthcare professionals should be more careful about the ethical implications of using photographs in publications given that many intersex persons describe their experience with medical photography as dehumanizing.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9596528/

Record 588

pmcid: PMC8923929

pmid: 35293180

doi: 10.7507/1002-1892.202111016

title: [Research progress of external volume expansion assisted autologous fat grafting for breast reconstruction].

authors

- Ma X
- Li F

journal: Zhongguo xiu fu chong jian wai ke za zhi = Zhongguo xiufu chongjian waike zazhi = Chinese journal of reparative and reconstructive surgery

journalAbbr: Zhongguo Xiu Fu Chong Jian Wai Ke Za Zhi

year: 2022

pubdate: 2022 Mar 15

volume: 36

issue: 3

pages: 370-375

abstract: OBJECTIVE: To review the application progress, mechanism, application points, limitations, and oncological safety of external volume expansion (EVE) assisted autologous fat grafting for breast reconstruction and provide a reference for optimizing the design of EVE. METHODS: Based on the latest relevant articles, the basic experiments and clinical applications of EVE were summarized. RESULTS: EVE can reduce interstitial fluid pressure, increase blood supply, and promote adipogenic differentiation, thereby benefiting the survival of transplanted fat. EVE assisted autologous fat grafting in clinical practice can improve the retention rate of breast volume and the outcome of breast reconstruction, meanwhile it doesn't increase the risk of local recurrence. But there is no standard parameters for application, and there are many complications and limitations. CONCLUSION: EVE improves the survival of transplanted fat, but its complications and poor compliance are obvious, so it is urgent to further investigate customized products for breast reconstruction after breast cancer and establish relevant application guidelines.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8923929/

Record 589

pmcid: PMC9379246

pmid: 35972647

doi: 10.1007/s00428-022-03379-4

title: 34(th) European Congress of Pathology - Abstracts.

authors

journal: Virchows Archiv : an international journal of pathology

journalAbbr: Virchows Arch

year: 2022

pubdate: 2022 Aug 16

volume: 481

issue: Suppl 1

pages: 1-364

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9379246/

Record 590

pmcid: PMC9801065

pmid: 36573035

doi: 10.12659/AJCR.937404

title: A 14-Year-Old Saudi Boy with Gynecomastia, Cushing Syndrome, Large-Cell Calcifying Sertoli Cell Tumor of the Testis, and Carney Complex.

authors

- Alruwaili TK
- Alkanhal KI
- AlShoomi AM
- Almanie DA
- Alahmed MH

journal: The American journal of case reports

journalAbbr: Am J Case Rep

year: 2022

pubdate: 2022 Dec 27

volume: 23

issue:

pages: e937404

abstract: BACKGROUND Carney complex (CNC) is a rare multiple neoplasia syndrome with autosomal dominant inheritance. CNC is frequently misdiagnosed owing to its diverse clinical characteristics. We reported the case of a 14-year-old Saudi boy with a history of gynecomastia, Cushing syndrome, large-cell calcifying Sertoli cell tumor of the testis, and CNC. CASE REPORT The patient was referred to the pediatric endocrine clinic for evaluation of bilateral slow progressing gynecomastia for 1-year duration. His clinical examination revealed lentigenes, bilateral diffuse breast enlargement (consistent with Tanner stage III), and asymmetrical testicular enlargement, more on the left side. Other systemic examinations were unremarkable. The initial blood workup showed elevated estradiol level with unsuppressed cortisol after an overnight 1-mg dexamethasone suppression test. Breast ultrasound (US) confirmed true gynecomastia. Testicular US revealed microcalcification and the testicular biopsy confirmed diagnoses of large-cell calcifying Sertoli cell tumor (LCCSCT). A 2-step dexamethasone suppression test showed a paradoxical rise in serum and urine cortisol levels, which are characteristic for PPNAD. LCCSCT and PPNAD are 2 major criteria fulfilling a diagnosis of CNC. The gene test showed heterozygous mutation in the PRKAR1A gene, which is diagnostic for CNC. The patient underwent bilateral mastoplasty and was planned for radical left orchiectomy. CONCLUSIONS Gynecomastia and LCCSCT can be presenting features of CNC, which mandates careful, thorough clinical examination and tailored investigation to reach a diagnosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9801065/

Record 591

pmcid: PMC10281649

pmid: 35675851

doi: 10.1016/j.ijrobp.2022.05.041

title: A Comparison of Patient- and Clinician-Reported Acute Toxic Effects During Radiation Therapy for Primary Breast Cancer.

authors

- Lapen K
- King C
- Braunstein LZ
- Khan AJ
- Kamrava MR
- Gillespie EF
- Cook KA

journal: International journal of radiation oncology, biology, physics

journalAbbr: Int J Radiat Oncol Biol Phys

year: 2022

pubdate: 2022 Oct 1

volume: 114

issue: 2

pages: 301-309

abstract: PURPOSE: Evidence suggests that cancer treatment-related toxic effects are underreported by

clinicians. We sought to compare patient- and clinician-reported acute toxic effects among patients undergoing radiation therapy for primary breast cancer and to determine factors associated with patient-clinician discordance. METHODS AND MATERIALS: Patient responses from a weekly Patient-Reported Outcomes version of the Common Terminology Criteria for Adverse Events based assessment were matched to clinician assessments of acute toxic effects during treatment. Weighted κ statistics were used to evaluate agreement between patient and clinician assessments. Linear regression, logistic regression, and generalized estimating equation models were used to identify covariates associated with discordance. RESULTS: Overall, 842 patient-clinician assessment pairs from 376 unique patients were analyzed. Total symptom burden score was higher for patients than clinicians (4.7 vs 2.3, P < .01). Dermatitis, pruritis, pain, and edema items were classified as having minimal agreement (κ of 0.25, 0.23, 0.20, and 0.25, respectively). Fatigue (κ 0.17) and psychosocial (0.03) patient-clinician pairs were found to have no agreement. The linear regression demonstrated that assessments by patients who identified as Black or African American were associated with a 0.13-point decrease in discordance (95% confidence interval, -0.25 to -0.01), while time from the start of treatment was associated with increased discordance (95% confidence interval, 0.07-0.12). CONCLUSIONS: For patients undergoing breast radiation therapy, discordance in patient and clinician symptom reporting is high and increases as treatment progresses. The mechanism of reduced discordance among Black or African American patients warrants further investigation. Prospective studies are needed to determine whether interventions for lower severity symptoms, which are commonly overlooked by clinicians, can reduce symptom burden and improve patient quality of life during radiation therapy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10281649/

Record 592

pmcid: PMC8950842

pmid: 35328752

doi: 10.3390/ijms23063332

title: Aberrant Notch Signaling Pathway as a Potential Mechanism of Central Precocious Puberty.

authors

- Shim YS
- Lee HS
- Hwang JS

journal: International journal of molecular sciences

journalAbbr: Int J Mol Sci

year: 2022

pubdate: 2022 Mar 19

volume: 23

issue: 6

pages:

abstract: The Notch signaling pathway is highly conserved during evolution. It has been well documented that Notch signaling regulates cell proliferation, migration, and death in the nervous, cardiac, and endocrine systems. The Notch pathway is relatively simple, but its activity is regulated by numerous complex mechanisms. Ligands bind to Notch receptors, inducing their activation and cleavage. Various post-translational processes regulate Notch signaling by affecting the synthesis, secretion, activation, and degradation of Notch pathway-related proteins. Through such post-translational regulatory processes, Notch signaling has versatile effects in many tissues, including the hypothalamus. Recently, several studies have reported that mutations in genes related to the Notch signaling pathway were found in patients with central precocious puberty (CPP). CPP is characterized by the early activation of the hypothalamus-pituitary-gonadal (HPG) axis. Although genetic factors play an important role in CPP development, few associated genetic variants have been identified. Aberrant Notch signaling may be associated with abnormal pubertal development. In this review, we discuss the current knowledge about the role of the Notch signaling pathway in puberty and consider the potential mechanisms

underlying CPP.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8950842/

Record 593

pmcid: PMC9067781

pmid:

doi: 10.4103/2230-8210.342222

title: Abstract 101: A case of CYP11A1 enzyme deficiency with unambiguity.

authors

- Vishwanath S
- Bantwal G
- Ayyar V
- George B
- Appaiah S

journal: Indian Journal of Endocrinology and Metabolism

journalAbbr: Indian J Endocrinol Metab

year: 2022

pubdate: 2022 Mar

volume: 26

issue: Suppl 1

pages: S37

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9067781/

Record 594

pmcid: PMC9067760

pmid:

doi: 10.4103/2230-8210.342251

title: Abstract 126: Myriad endocrine manifestations of a rare syndrome.

authors

- Togarla S
- Sahay R
- Neelaveni

journal: Indian Journal of Endocrinology and Metabolism

journalAbbr: Indian J Endocrinol Metab

year: 2022

pubdate: 2022 Mar

volume: 26

issue: Suppl 1

pages: S43-4

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9067760/

Record 595

pmcid: PMC9494561

pmid: 36130823

doi: 10.1136/bcr-2022-250444

title: Adult-onset reversible idiopathic hypogonadotropic hypogonadism in male adult carrying a WDR11 missense mutation.

authors

- Yamada R
- Yamakita N
- Yasuda K
- Imai A

journal: BMJ case reports

journalAbbr: BMJ Case Rep

year: 2022

pubdate: 2022 Sep 21

volume: 15

issue: 9

pages:

abstract: Idiopathic hypogonadotropic hypogonadism (IHH) occurs mostly in childhood or adolescence and very rarely in adulthood. It is characterised by delayed onset of secondary sexual characteristics. Many genetic abnormalities have been reported in congenital IHH cases, but rarely in adult-onset IHH cases. IHH requires lifelong hormone replacement therapy; however, a few reports suggest the reversibility of this condition.In this case, after having his first child, a man in his 20s was diagnosed with gynecomastia followed by IHH. He improved with gonadotropin-releasing hormone replacement therapy and had two more children. The treatment was discontinued after 4 years, but the improvement was sustained. He had a heterozygous missense variant in WDR11 (c.2390G>A; p.Arg797His), which may play a role in adult-onset IHH reversal. Accumulation of such cases can contribute to our understanding of the pathogenesis and genetic component of IHH.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9494561/

Record 596

pmcid: PMC9483724

pmid: 36133826

doi: 10.3389/fphar.2022.923939

title: Adverse drug reactions and correlations with drug-drug interactions: A retrospective study of reports from 2011 to 2020.

authors

- Jiang H
- Lin Y
- Ren W
- Fang Z
- Liu Y

- Tan X
- Lv X
- Zhang N

journal: Frontiers in pharmacology

journalAbbr: Front Pharmacol

year: 2022

pubdate: 2022

volume: 13

issue:

pages: 923939

abstract: Introduction: Adverse drug reactions (ADRs) represent a public health problem worldwide that deserves attention due to the impact on mortality, morbidity, and healthcare costs. Drug-drug interactions (DDIs) are an important contributor to ADRs. Most of the studies focused only on potential DDIs (pDDIs), while the detailed data are limited regarding the ADRs associated with actual DDIs. Methods: This retrospective study evaluated ADRs reported between 2011 and 2020 in a tertiary hospital. The causality and severity of ADRs were evaluated through the Naranjo Algorithm and Hartwig's scale, respectively. Preventability classification was based on the modified Schoumock and Thornton scale. For ADRs with at least two suspected drugs, pDDIs were identified according to the Lexi-Interact. We further checked whether the ADR description in the reports corresponded to the clinical consequences of the pDDIs. Results: A total of 1,803 ADRs were reported, of which 36.77% ADRs were classified as mild, 43.26% as moderate, and 19.97% as severe. The assessment of causality showed that the distributions of definite, probable, and possible categories were 0.33%, 58.68%, and 40.99%, respectively. A total of 53.97% of ADRs were identified as preventable ADRs, while 46.03% were recognized as unpreventable. The severity of ADRs was significantly correlated with age, the number of suspected drugs and preventability. Antimicrobial agents were the most common implicated pharmacological group, and the most frequently affected system was the gastrointestinal system. Considering individual drugs, aspirin was the most frequently reported drug. Among 573 ADRs with at least two suspected drugs, 105 ADRs were caused by actual DDIs, of which only 59 and 6 ADRs were caused by actual DDIs in category D and X, respectively. The most frequent drugs involved in actual DDIs of category D were aspirin and heparin, with the majority of ADRs being gastrointestinal bleeding. Conclusion: This study analyzed the pattern of ADRs in detail and obtained clinical evidence about ADRs associated with actual DDIs. These findings may be useful to compare patterns between different centers and to design preventive strategies for ADRs. Continuous education and training should be provided for physicians regarding the knowledge and recognition of ADRs associated with DDIs.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9483724/

Record 597

pmcid: PMC9577820

pmid: 36268378

doi: 10.1016/j.amsu.2022.104720

title: An Indonesian male with congenital hypogonadotropic hypogonadism: A case report and literature review.

authors

- Arrosy LS
- Novida H

journal: Annals of medicine and surgery (2012)

journalAbbr: Ann Med Surg (Lond)

year: 2022

pubdate: 2022 Oct

volume: 82

issue:

pages: 104720

abstract: INTRODUCTION: Congenital hypogonadotropic hypogonadism (CHH) is a rare disorder caused by insufficient gonadotropin-releasing hormone (GnRH) production. CASE PRESENTATION: An Indonesian adolescent, 22 years old, Javanese ethnic, complained of a small penis, low sexual desire, fatigue, and anosmia since childhood. Medical history stated that the patient had low testosterone levels 7 years ago and received testosterone once. Testosterone, luteinizing hormone (LH), and follicle-stimulating hormone (FSH) were decreased. The testicular ultrasound result was bilateral microtestis, suspicious of bilateral hypoplasia of the epididymis. Brain MRI also supports the diagnosis of hypogonadotropic hypogonadism, and the patient received Sustanon of 250 mg/2 weeks. The patient showed a good prognosis after 1 month of therapy. DISCUSSION: The success of CHH therapy must be explored to improve its management. CONCLUSION: CHH in an Indonesian male shows a good prognosis with testosterone injection.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9577820/

Record 598

pmcid: PMC9323942

pmid: 35885460

doi: 10.3390/diagnostics12071554

title: An Update on the General Features of Breast Cancer in Male Patients-A Literature Review.

authors

- Ionescu S
- Nicolescu AC
- Marincas M
- Madge OL
- Simion L

journal: Diagnostics (Basel, Switzerland)

journalAbbr: Diagnostics (Basel)

year: 2022

pubdate: 2022 Jun 26

volume: 12

issue: 7

pages:

abstract: Male breast cancers are uncommon, as men account for less than 1 percent of all breast carcinomas. Among the predisposing risk factors for male breast cancer, the following appear to be significant: (a) breast/chest radiation exposure, (b) estrogen use, diseases associated with hyper-estrogenism, such as cirrhosis or Klinefelter syndrome, and (c) family health history. Furthermore, there are clear familial tendencies, with a higher incidence among men who have a large number of female relatives with breast cancer and (d) major inheritance susceptibility. Moreover, in families with BRCA mutations, there is an increased risk of male breast cancer, although the risk appears to be greater with inherited BRCA2 mutations than with inherited BRCA1 mutations. Due to diagnostic delays, male breast cancer is more likely to present at an advanced stage. A core biopsy or a fine needle aspiration must be performed to confirm suspicious findings. Infiltrating ductal cancer is the most prevalent form of male breast cancer, while invasive lobular carcinoma is extremely uncommon. Male breast cancer is almost always positive for hormone receptors. A worse prognosis is associated with a more advanced stage at diagnosis for men with breast cancer. Randomized controlled trials which recruit both female and male patients should be developed in order to gain more consistent data on the optimal clinical approach.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9323942/

Record 599

pmcid: PMC8997146

pmid: 35406575

doi: 10.3390/cancers14071803

title: Androgen Deprivation Therapy in High-Risk Localized and Locally Advanced Prostate Cancer.

authors

- Iwamoto H
- Izumi K
- Makino T
- Mizokami A

journal: Cancers

journalAbbr: Cancers (Basel)

year: 2022

pubdate: 2022 Apr 1

volume: 14

issue: 7

pages:

abstract: The recommended treatment for high-risk localized or locally advanced prostate cancer is radical prostatectomy plus extended pelvic lymph node dissection or radiation therapy plus long-term androgen deprivation therapy. However, some patients are treated with androgen deprivation therapy alone for various reasons. In this review, we will discuss the position, indications, complications, and future prospects of androgen deprivation therapy for high-risk localized and locally advanced prostate cancer.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8997146/

Record 600

pmcid: PMC9297041

pmid: 35874458

doi: 10.2147/CCID.S289750

title: Androgen Receptor Inhibitors in the Treatment of Acne Vulgaris: Efficacy and Safety Profiles of Clascoterone 1% Cream.

authors

- Sanchez C
- Keri J

journal: Clinical, cosmetic and investigational dermatology

journalAbbr: Clin Cosmet Investig Dermatol

year: 2022

pubdate: 2022

volume: 15

issue:

pages: 1357-1366

abstract: The purpose of this narrative review is to provide a summary of the clinical trials on the efficacy and safety of clascoterone 1% cream (Winlevi) to grant providers an understanding of which patients will benefit most from this novel topical antiandrogen medication. Clascoterone 1% cream (Winlevi) offers a new and exciting treatment approach for a difficult and common skin condition such as acne vulgaris. This topical androgen antagonist is the first of its kind but will hopefully provoke investigations into other androgen receptor antagonists with similar or better efficacy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9297041/

Record 601

pmcid: PMC9497988

pmid: 36140569

doi: 10.3390/diagnostics12092168

title: Approach of Heterogeneous Spectrum Involving 3beta-Hydroxysteroid Dehydrogenase 2 Deficiency.

authors

- Nicola AG
- Carsote M
- Gheorghe AM
- Petrova E
- Popescu AD
- Staicu AN
- Țuculină MJ
- Petcu C
- Dascălu IT
- Tircă T

journal: Diagnostics (Basel, Switzerland)

journalAbbr: Diagnostics (Basel)

year: 2022

pubdate: 2022 Sep 7

volume: 12

issue: 9

pages:

abstract: We aim to review data on 3beta-hydroxysteroid dehydrogenase type II (3βHSD2) deficiency. We identified 30 studies within the last decade on PubMed: 1 longitudinal study (N = 14), 2 cross-sectional studies, 1 retrospective study (N = 16), and 26 case reports (total: 98 individuals). Regarding geographic area: Algeria (N = 14), Turkey (N = 31), China (2 case reports), Morocco (2 sisters), Anatolia (6 cases), and Italy (N = 1). Patients' age varied from first days of life to puberty; the oldest was of 34 y. Majority forms displayed were salt-wasting (SW); some associated disorders of sexual development (DSD) were attendant also-mostly 46,XY males and mild virilisation in some 46,XX females. SW pushed forward an early diagnosis due to severity of SW crisis. The clinical spectrum goes to: premature puberty (80%); 9 with testicular adrenal rest tumours (TARTs); one female with ovarian adrenal rest tumours (OARTs), and some cases with adrenal hyperplasia; cardio-metabolic complications, including iatrogenic Cushing' syndrome. More incidental (unusual) associations include: 1 subject with Barter syndrome, 1 Addison's disease, 2 subjects of Klinefelter syndrome (47,XXY/46,XX, respective 47,XXY). Neonatal screening for 21OHD was the scenario of detection in some cases; 17OHP might be elevated due to peripheral production (pitfall for misdiagnosis of 21OHD). An ACTH stimulation test was used in 2 studies. Liquid chromatography tandem-mass spectrometry unequivocally sustains the diagnostic by expressing high baseline 17OH-pregnenolone to cortisol ratio as well as 11-

oxyandrogen levels. HSD3B2 gene sequencing was provided in 26 articles; around 20 mutations were described as "novel pathogenic mutation" (frameshift, missense or nonsense); many subjects had a consanguineous background. The current COVID-19 pandemic showed that CAH-associated chronic adrenal insufficiency is at higher risk. Non-adherence to hormonal replacement contributed to TARTs growth, thus making them surgery candidates. To our knowledge, this is the largest study on published cases strictly concerning 3βHSD2 deficiency according to our methodology. Adequate case management underlines the recent shift from evidence-based medicine to individualized (patient-oriented) medicine, this approach being particularly applicable in this exceptional and challenging disorder.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9497988/

Record 602

pmcid: PMC9514273

pmid: 36177431

doi: 10.4103/jehp.jehp_311_22

title: Assessment of knee and ankle proprioception during the third trimester of pregnancy and postpartum period among primiparous women: An observational longitudinal study.

authors

- Wakkar A
- Patil SP

journal: Journal of education and health promotion

journalAbbr: J Educ Health Promot

year: 2022

pubdate: 2022

volume: 11

issue:

pages: 241

abstract: BACKGROUND: Pregnant women experience falls, particularly in the third trimester. In this population, physiological changes, as well as ligament laxity, might influence joint proprioception and do not return to normal during the postpartum period. The prevalence of falls during pregnancy and postpartum periods imposes a need to study proprioception in pregnant women and the postpartum period. MATERIALS AND METHOD: An observational longitudinal study was conducted in June 2018 in outpatient clinic Chinchwad Pune. A total of 36 primiparous women were included in the study by using purposive sampling. The mean and standard deviation of the age was 25.92 (2.59). Proprioception was assessed for the knee joint and the ankle joint during the third trimester of pregnancy and 6 th and 12 th week postpartum. Outcomes included were the Joint Reposition Test for both knee and ankle joints using UTHSCSA Image Tool Software 3.0. Repeated-measure ANOVA was performed for the normally distributed data, and nonparametric test Friedman's test was performed for data that were not distributed normally. The data was statistically analyzed using the SPSS software version 26. The level of significance was set at 0.05, confidence intervals of 95% were used. RESULT: The result revealed significant ($P < 0.05$) improvement in both knee and ankle proprioception during the third trimester of pregnancy and postpartum period 6 th and 12 th week. CONCLUSION: Knee proprioception and ankle proprioception were found to improve significantly during the postpartum period 6 th and 12 th weeks compared to the third trimester of pregnancy but do not return to the prepregnancy state.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9514273/

Record 603

pmcid: PMC9993321

pmid: 36910346

doi: 10.4314/ahs.v22i3.17

title: Association of ATM, CDH1 and TP53 genes polymorphisms with familial breast cancer in patients of Khyber Pakhtunkhwa, Pakistan.

authors

- Rahim A
- Zakiullah
- Jan A
- Ali J
- Khuda F
- Muhammad B
- Khan H
- Shah H
- Akbar R

journal: African health sciences

journalAbbr: Afr Health Sci

year: 2022

pubdate: 2022 Sep

volume: 22

issue: 3

pages: 145-154

abstract: BACKGROUND: Genetic studies play a significant role in understanding the underlying risk factors of breast cancer. Polymorphism in the tumor suppressor gene TP 53, CDH1 and ATM genes are found to increase susceptibility for breast cancer globally. OBJECTIVE: This study aimed to identify/analyze the contribution of genetic polymorphisms in the breast cancer candidate genes ATM, TP53 and CDH1 that may be associated with familial breast cancer risk in the Khyber Pakhtunkhwa population. SUBJECTS AND METHODS: In the present case-control study, Whole Exome Sequencing (WES) of the 100 breast cancer patients and 100 ethnic controls were performed for the selected genes in the target population. RESULTS: Of the studied variants rs3743674 of the CDH1 gene (crude P=0.014 and adjusted p=0.000) evident significant association with breast cancer in Pakistani Pashtun population. Whereas TP53rs1042522 (crude P=0.251 and adjusted P=0.851) and ATM rs659243 (crude p=0.256 and adjusted p=0.975) showed no or negative association with breast cancer in study population. CONCLUSION: The present study demonstrates that CDH1rs3743674 polymorphism is associated with elevated breast cancer risk in the Pashtun ethic population of Khyber Pakhtunkhwa.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9993321/

Record 604

pmcid: PMC8956902

pmid: 35382047

doi: 10.11604/pamj.2022.41.31.30195

title: Autoimmune polyglandular syndrome type 2 in an HIV-positive man managed at the University Teaching Hospitals Lusaka, Zambia: a case report.

authors

- Meek M
- Kabengele C
- Kamanga B

journal: The Pan African medical journal

journalAbbr: Pan Afr Med J

year: 2022

pubdate: 2022

volume: 41

issue:

pages: 31

abstract: Autoimmune polyglandular syndromes (APS) are rare autoimmune endocrinopathies characterized by the coexistence of at least two endocrine gland insufficiencies developed from autoimmune mechanisms. APS may also be associated with non-endocrine immune diseases. In HIV infection, antiretroviral therapy can improve the quality of life to reduce the incidence of opportunistic infections, malignancies, and death. HIV disease may also be associated with complications, such as immune reconstitution inflammatory syndrome (IRIS) presenting as infections, malignancies or autoimmune diseases. We here report the clinical case of an HIV-infected man receiving antiretroviral therapy, who subsequently developed APS type II, characterized by Grave's disease, type 1 diabetes mellitus. He complained of a mass in his anterior neck, diarrhea, weight loss, palpitations, hand tremors and excessive sweating. Six months before he had been diagnosed with type 1 diabetes mellitus. The patient had a diffusely enlarged thyroid on ultrasound, elevated random blood glucose of 14.0 mmol/l; elevated free T4 at 5.03 ng/dL and suppressed thyroid-stimulating hormone (TSH) at <0.05 micro-IU/mL. The patient was treated with carbimazole and propranolol for Graves' thyrotoxicosis and basal bolus insulin regimen (actrapid and protaphane) for hyperglycemia. At monthly follow-up assessments he was euthyroid and 2-hour postprandial blood glucose test was normal. The goitre had markedly reduced in size. This screening for APS in HIV patients with autoimmune IRIS as well as patients with autoimmune endocrinopathies in order to allow for early diagnosis and prompt initiation of treatment to reduce the risk of morbidity and mortality.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8956902/

Record 605

pmcid: PMC9428391

pmid: 36059343

doi: 10.7759/cureus.27567

title: Bilateral Phyllodes in a 12-Year-Old Adolescent: Report of a Rare Case.

authors

- Kenjale SH
- Dighe SP Sr
- Shinde RK
- Jogdand SD
- Ghode DB

journal: Cureus

journalAbbr: Cureus

year: 2022

pubdate: 2022 Aug

volume: 14

issue: 8

pages: e27567

abstract: Phyllodes tumors, previously known as cystosarcoma phyllodes, are fibroepithelial tumors that consist

of epithelial and cellular stromal components. These tumors have a predilection to recur after wide local excision and to attain massive sizes. They account for less than 0.5% of cases of all breast neoplasms and are primarily found in the third to fourth decade of life, and rarely in adolescence. However, less than 25 cases are reported to date wherein this tumor is found in adolescent females, with this case being one of those. This is a report of a rare case of a 12-year-old premenarchal female with bilateral phyllodes tumor, highlighting its peculiarities, diagnostic features, and surgical management in view of the patient's age and quality of life.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9428391/

Record 606

pmcid: PMC9313844

pmid: 35357024

doi: 10.1002/pros.24328

title: Bipolar androgen therapy (BAT): A patient's guide.

authors

- Denmeade S
- Antonarakis ES
- Markowski MC

journal: The Prostate

journalAbbr: Prostate

year: 2022

pubdate: 2022 May

volume: 82

issue: 7

pages: 753-762

abstract: Bipolar androgen therapy (BAT) is a new treatment concept for men whose prostate cancer has become resistant to standard hormone-blocking therapy. Over the past decade, we have performed a series of clinical studies testing BAT in asymptomatic men with castration-resistant prostate cancer. The key findings from these clinical studies are that BAT (a) can be safely administered to asymptomatic patients with metastatic castrate-resistant prostate cancer; (b) does not produce symptomatic disease progression; (c) produces sustained prostate-specific antigen and objective responses in 30%-40% of patients; and (d) can resensitize and prolong response to subsequent antiandrogen therapy. The concept of BAT has generated significant interest from men with prostate cancer, their families, and their physicians. Here we provide a "Patient's Guide" that answers questions about BAT in a form that is accessible to patients, their families, and physicians. Our goal is to provide information to help patients make the most informed decisions they can regarding their prostate cancer treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9313844/

Record 607

pmcid: PMC9577873

pmid: 36268300

doi: 10.1016/j.amsu.2022.104774

title: Breast abscess in two Nepalese newborns: An unusual series.

authors

- Shrestha AL
- Mishra A

journal: Annals of medicine and surgery (2012)

journalAbbr: Ann Med Surg (Lond)

year: 2022

pubdate: 2022 Oct

volume: 82

issue:

pages: 104774

abstract: BACKGROUND: Breast abscess in newborns is an exceedingly rare pyogenic inflammation that usually starts as mastitis neonatorum. Although mastitis can respond to antibiotic therapy in the initial stages, once advanced into a purulent collection, decompression may be indicated either in the form of needle aspiration or even surgical drainage. We present two newborns with breast abscesses managed surgically with successful outcomes. CASE PRESENTATION: Case 1: A 13-day-old boy presented with swelling and redness over the left breast for a day. Local examination revealed a warm, indurated swelling with redness and fluctuation. Needle aspiration confirmed pus. Intravenous (IV) antibiotics were started right away followed by pus drainage, the culture of which yielded Methicillin-Sensitive Staphylococcus aureus (MSSA). Regular wound care subsequently resulted in complete recovery.Case 2: A 15-day-old boy presented with swelling, redness and milky discharge from the right breast for two days. The swelling was erythematous, tender and indurated but not cystic or fluctuant. Ultrasonogram was suggestive of an abscess. Once again, IV antibiotics were administered followed by drainage. Pus yielded MSSA. Following regular wound dressings, satisfactory healing was achieved. CONCLUSION: Neonatal mastitis and breast abscess are uncommon. Early recognition followed by appropriate antibiotic therapy and drainage of the abscess is the mainstay of treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9577873/

Record 608

pmcid: PMC9453666

pmid: 36156909

doi: 10.1159/000522616

title: Breast Augmentation with Autologous Fat Grafting Immediately after Removal of Polyacrylamide Hydrogel and Fibrotic Capsule in 162 Patients.

authors

- Gao Q
- Zhai P
- Qi J
- Yang Z
- Hu Y
- Yuan X
- Liu C
- Qi Z

journal: Breast care (Basel, Switzerland)

journalAbbr: Breast Care (Basel)

year: 2022

pubdate: 2022 Aug

volume: 17

issue: 4

pages: 377-384

abstract: OBJECTIVE: In this study, we investigated the feasibility and efficacy of immediate breast augmentation with autologous fat grafting after removal of polyacrylamide hydrogel (PAAG) and fibrotic capsule. METHODS: A retrospective study was conducted on 162 female patients who underwent removal of breast filler PAAG and the fibrotic capsule which produced after injection of PAAG via areola omega-shaped incision. Then autologous fat grafting was immediately performed evenly and radially around the areola via the same incision into different layers (subcutaneous, submammary tissue, pectoralis major intramuscular, and inferior pectoralis major space) except the empty cavity. The cavity left by removal of PAAG and fibrous capsule was closed with negative pressure drainage tube and slight external pressure. RESULTS: All patients recovered well without severe complications. The average score of postoperative satisfaction with physical well-being: chest was 99.83 (total score: 100) compared with the average satisfaction score of 71.69 (total score: 100) preoperatively by means of BREAST-Q™ evaluation ($p < 0.01$). All patients were satisfied with their postoperative breast shape. CONCLUSIONS: Removing as much as possible is critical for patients who underwent the PAAG injection. Our experience in immediate breast augmentation with autologous fat grafting after removal of PAAG and fibrotic capsule proved useful and effective to maintain the balance between removing the PAAG as much as possible and retaining soft tissue to reshape breasts. LEVEL OF EVIDENCE: IV.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9453666/

Record 609

pmcid: PMC9779403

pmid: 36554776

doi: 10.3390/ijerph192416895

title: Breast Fluctuating Asymmetry in Women with Macromastia/Gigantomastia.

authors

- Kasielska-Trojan A
- Zawadzki T
- Antoszewski B

journal: International journal of environmental research and public health

journalAbbr: Int J Environ Res Public Health

year: 2022

pubdate: 2022 Dec 16

volume: 19

issue: 24

pages:

abstract: BACKGROUND: A number of studies have reported breasts have high fluctuating asymmetry (FA:|Right-Left|), suggesting that they contain evolutionary and clinical information (e.g., high FA correlates with breast cancer risk). Here we focus on breast FA in women with a wide range of breast sizes, including participants with macromastia and/or gigantomastia. MATERIAL AND METHODS: The sample included 65 women (mean age 33.97 ± 12.1 years). Thirty were randomly selected students and/or patients who regarded their breast size as small, normal or average and who had not undergone or intended to have any breast surgery. The remainder ($n = 35$) were qualified for breast reduction due to macromastia and/or gigantomastia. In all participants we measured/calculated weight, height and BMI, as well as took chest photographs. Breast volumes and nipple areola complex FAs were evaluated in a specifically-designed software. RESULTS: Breast size significantly positively correlated with breast volume FA in all women ($t = 5.17$, $p < 0.0001$) and in women with macromastia/gigantomastia ($t = 2.32$, $p = 0.027$). All nipple location FAs correlated positively with breast size. CONCLUSIONS: In women with different breast sizes, breast size correlates with FA calculated from breast volume and nipple location FAs. In women with macromastia and/or gigantomastia, breasts present higher FA than "normal" breasts. This observation may give a rationale for earlier and more frequent

prophylactic breast imaging in women with macromastia and/or gigantomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9779403/

Record 610

pmcid: PMC9434022

pmid: 36030763

doi: 10.1016/j.ijscr.2022.107520

title: Breast Implant-Associated Anaplastic Large Cell Lymphoma (BIA-ALCL) in a young transgender woman: A case report.

authors

- Materazzo M
- Vanni G
- Rho M
- Buonomo C
- Morra E
- Mori S

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2022

pubdate: 2022 Sep

volume: 98

issue:

pages: 107520

abstract: INTRODUCTION: Breast Implant-Associated Anaplastic Large Cell Lymphoma (BIA-ALCL) is a rare T-cell lymphoma occurring after breast implant procedures. As gender confirmation therapy (GCT) in male-to-female transgender (FT), up to 60-70 % of patients require breast augmentation and are at risk for BIA-ALCL. Hence, we report the youngest BIA-ALCL case in the Italian population and the first early-stage BIA-ALCL occurred in FT patients. CASE PRESENTATION: A 27-years-old FT was admitted to outpatients' clinics due to swollen left breast. The patient underwent GCT with a macrotextured implant four years before. Clinical examination revealed swollen left breast. Ultrasound and magnetic resonance imaging confirmed left breast periprosthetic effusion. Positron emission tomography-computed tomography scan did not reveal any focal pathological uptake. Fine needle aspiration cytology confirmed BIA-ALCL suspect. The patient underwent bilateral en bloc breast implant removal and periprosthetic capsulectomy. Due to the early stage, adjuvant chemotherapy was omitted. Postoperative follow-up was unremarkable. CLINICAL DISCUSSION: BIA-ALCL is a rare, emergent clinical concern after breast implant surgery. GCT leads to improved body satisfaction and quality of life in FT individuals. As for non-trans patients undergoing breast reconstruction or breast augmentation, this clinical case once again demonstrates that FT patients undergoing breast implant surgery are at risk of BIA-ALCL. CONCLUSION: Physicians should promote awareness among patients' GCT and tailored postoperative follow-up.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9434022/

Record 611

pmcid: PMC8797826

pmid: 35178219

doi: 10.22088/cjim.13.1.132

title: Breast metastasis from gastric carcinoma: A case report.

authors

- Kiani Markani L
- Kiani Markani F
- Kadivar M
- Hossinaei N
- Safari E

journal: Caspian journal of internal medicine

journalAbbr: Caspian J Intern Med

year: 2022

pubdate: 2022 Winter

volume: 13

issue: 1

pages: 132-135

abstract: BACKGROUND: There are rare cases of breast metastasis from gastric carcinoma origin. In this regard, we presented a case of signet ring cell stomach cancer with metastasis to the breast. CASE PRESENTATION: The case was a 44-year-old female with a history of gastric cancer and chief complaint of progressive bilateral and gradual breast enlargement and mass palpation 6 years after stomach surgery. An excisional biopsy of the right breast was performed at the early phase of her clinical symptoms with the pathology at the early phase of fibroadenoma and sclerosing adenosis. Given the persistent right breast thickening and enlargement, the ultrasonography and MRI together showed right breast large masses, ductal enhancement in left retro areolar space beside the bilaterally enlarged axillary lymph nodes after 6 months. In this phase, the core needle biopsy showed right breast mass adenocarcinoma consistently with metastatic gastric carcinoma and also core needle biopsy of both axillary lymph nodes indicating the involvement by tumor consistently with metastatic gastric carcinoma. In the IHC, breast tumor cells were negative for ER, PR, HER2/neu, GATA3, GCDFP15 and CK20, but positive for CK7, CK19, and CDx2. CONCLUSION: The diagnosis of breast metastasis of gastric carcinoma was confirmed according to the past history of patient, histological finding, and immune-histological markers.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8797826/

Record 612

pmcid: PMC8918540

pmid: 35295983

doi: 10.3389/fendo.2022.810375

title: Case Report: A Novel Point Mutation of SOX3 in a Subject With Growth Hormone Deficiency, Hypogonadotrophic Hypogonadism, and Borderline Intellectual Disability.

authors

- Li J
- Zhong Y
- Guo T
- Yu Y
- Li J

journal: Frontiers in endocrinology

journalAbbr: Front Endocrinol (Lausanne)

year: 2022

pubdate: 2022

volume: 13

issue:

pages: 810375

abstract: SOX3 is critical for the development of the pituitary, brain, and face, and SOX3 mutations may lead to hypopituitarism, intellectual disability, and craniofacial abnormalities. Common SOX3 mutations are duplications and deletions of the whole or part of SOX3 , yet only a few cases with point mutations were reported by far. We present a case with growth retardation, small penis, and learning difficulty. Further assessment confirmed growth hormone deficiency, hypogonadotropic hypogonadism (HH), and borderline intellectual disability. He also responded well to gonadotropin-releasing hormone stimulation test, which suggests defects in the hypothalamus, contrary to previous studies that reported defects in the pituitary. A pathogenic frame-shift mutation of SOX3 was found. A heterogeneous missense mutation in SEMA3A was identified in this patient as well, which may also contribute to the development of HH. As far as we know, this is the first report that a frame-shift mutation of SOX3 constitutes rare genetic causes of HH and growth hormone deficiency. Whether mutations in these two genes act synergistically in the pathogenesis of the patient's phenotype remains to be further investigated. We believe that our case extends the phenotypic spectrum and genetic variability of SOX3 mutation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8918540/

Record 613

pmcid: PMC9641066

pmid: 36386264

doi: 10.21037/tau-22-92

title: Case report: a study of the clinical characteristics and genetic variants of post-finasteride syndrome patients.

authors

- Li X
- Guo Y
- Lu Y
- Li H
- Yan S
- Li H
- Li Y

journal: Translational andrology and urology

journalAbbr: Transl Androl Urol

year: 2022

pubdate: 2022 Oct

volume: 11

issue: 10

pages: 1452-1457

abstract: BACKGROUND: Finasteride is widely used in the treatment of benign prostatic hyperplasia (BPH) and androgenic alopecia (AGA). Post-finasteride syndrome (PFS) is a spectrum of persistent symptoms reported by some patients after treatment with finasteride for androgenetic alopecia. These patients show many abnormal clinical manifestations, including psychological disorders (depression and anxiety, among others) and sexual dysfunction. However, there is insufficient research on the persistent severe side effects in young male patients with PFS, and the underlying mechanism of PFS has not been fully elucidated. Growing evidence highlights the relevance of genetic variants and their associated responses to drugs. Therefore, we performed next-generation

sequencing (NGS) in our study of PFS. CASE DESCRIPTION: Here, we enrolled three young male patients aged 20-30 years with a PFS duration of 1-3 years and analyzed their clinical and genetic information. PFS patients suffered from erectile dysfunction (ED), anxiety, feelings of isolation, and insomnia. Variants in genes, including CA8 , VSIG10L2 , HLA-B , KRT38 and HLA-DRB1 , were detected, and these genes represent potential risk genes. CONCLUSIONS: PFS, commonly observed in young men, has certain clinical manifestations, mainly psychological disorders and abnormal sexual functions. Young men who may take finasteride therapy for hair loss should receive consultation services and be informed of possible future harms. Psychological screening is an important method to reduce the occurrence of PFS. At present, the underlying mechanism of PFS is not very clear, and more research is needed to improve the understanding of the disease. Some genes are abnormal in PFS patients, suggesting that clinical and genetic evaluation might be needed before the prescription of 5α-reductase inhibitors. With research progress, genetic screening may be a promising way to avoid the harmful effects of finasteride in people with related genetic risk factors.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9641066/

Record 614

pmcid: PMC9499430

pmid: 36177259

doi: 10.1259/bjrcr.20210181

title: Child gigantomastia revealing juvenile giant fibroadenomas.

authors

- Belkouchi L
- El Haddad S
- Mrani Alaoui N
- Allali N
- Chat L

journal: BJR case reports

journalAbbr: BJR Case Rep

year: 2022

pubdate: 2022 Mar 10

volume: 8

issue: 2

pages: 20210181

abstract: Breast masses in children and adolescents are uncommon. They can be caused by tumors such as fibroadenomas and phyllode tumors. These masses can cause gigantomastia, due to their rapidly increasing size. We report the case of a 12- year-old patient admitted in our department for a rapidly growing gigantomastia evolving in a matter of 8 months. Imaging features were in favor of juvenile giant fibroadenomas and diagnosis was confirmed by biopsy. Juvenile giant fibroadenomas are rare and represent 0.5-2% of all fibroadenomas, their exact etiology is unknown. They affect children and adolescents, with a predominance in African-American females. They may cause breast enlargement and asymmetry. The rapid growth causes anxiety and is the main cause of consultation. These tumors although benign, have to be treated rapidly because they can destruct up to 80% of the surrounding normal breast tissue, and conservatory treatment cannot be done.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9499430/

Record 615

pmcid: PMC9577501

pmid: 36277256

doi: 10.1093/schizbullopen/sgac059

title: Chronic Use of Antipsychotics in Schizophrenia: Are We Asking the Right Question?

authors

- Rubio JM
- Perez-Rodriguez M

journal: Schizophrenia bulletin open

journalAbbr: Schizophr Bull Open

year: 2022

pubdate: 2022 Jan

volume: 3

issue: 1

pages: sgac059

abstract: There is an ongoing debate about the potential risks and benefits of long-term antipsychotic treatment in schizophrenia. The data for and against the chronic use of these medicines is mostly indirect, either from observational studies potentially exposed to reverse causation bias or randomized controlled studies that do not cover beyond 2-3 years. We propose that perseverating on the question of what positive or negative outcomes are causally associated with chronic antipsychotic treatment may not lead to better answers than the limited ones that we have, given the limited feasibility of more conclusive studies. Rather, we argue that addressing the research question of the risks and benefits of antipsychotic discontinuation from a perspective of personalized medicine, can be more productive and meaningful to people living with schizophrenia. To this end, research that can quantify the risk of relapse after treatment continuation for a given individual should be prioritized. We make the case that clinically feasible neuroimaging biomarkers have demonstrated promise in related paradigms, and that could be offering a way past this long debate on the risks and benefits of chronic antipsychotic use.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9577501/

Record 616

pmcid: PMC9664805

pmid: 36376972

doi: 10.1186/s13256-022-03633-y

title: Clinical characterizations of three adults with genetically confirmed spinal muscular atrophy: a case series.

authors

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journal: Journal of medical case reports

journalAbbr: J Med Case Rep

year: 2022

pubdate: 2022 Nov 15

volume: 16

issue: 1

pages: 435

abstract: BACKGROUND: Spinal muscular atrophy is a recessively inherited autosomal neuromuscular disorder, with characteristic progressive muscle weakness. Most spinal muscular atrophy cases clinically manifest during infancy or childhood, although it may first manifest in adulthood. Although spinal muscular atrophy has come to the era of newborn screening and promising treatments, genetically confirmed spinal muscular atrophy patients are still rare in third world countries, including Indonesia. CASE PRESENTATIONS: We presented three Indonesian patients with spinal muscular atrophy genetically confirmed during adulthood. The first case was a 40-year-old male who presented with weakness in his lower limbs that started when he was 9 years old. At the age of 16 years, he could no longer walk and started using a wheelchair. He first came to our clinic at the age of 38 years, and was diagnosed with spinal muscular atrophy 2 years later. The second patient was a 58-year-old male who presented with lower limb weakness since he was 12 years old. Owing to the geographical distance and financial problems, he was referred to our clinic at the age of 56 years, when he already used a walker to walk. Lastly, the third patient was a 28-year-old woman, who was in the first semester of her second pregnancy, and who presented with slowly progressing lower limb weakness. Her limb weakness began at the age of 8 years, and slowly progressed until she became dependent on her wheelchair 8 years later until now. She had successfully given birth to a healthy daughter 3 years before her first visit to our clinic. All three patients were diagnosed with neuromuscular disorder diseases, with the differential diagnoses of Duchenne muscular dystrophy, spinal muscular atrophy, and Becker muscular dystrophy. These patients were finally confirmed to have spinal muscular atrophy due to SMN1 deletion by polymerase chain reaction and restriction fragment length polymorphism. CONCLUSIONS: Many genetic diseases are often neglected in developing countries owing to the difficulty in diagnosis and unavailable treatment. Our case series focused on the disease courses, diagnosis difficulties, and clinical presentations of three patients that finally lead to diagnoses of spinal muscular atrophy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9664805/

Record 617

pmcid: PMC8866913

pmid: 35228933

doi: 10.7759/cureus.21574

title: Clinical Spectrum and Causes of Delayed Puberty Among Patients Presenting to the Endocrine Clinic at Jinnah Postgraduate Medical Centre.

authors

- Zahra F
- Ahsan T
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journal: Cureus

journalAbbr: Cureus

year: 2022

pubdate: 2022 Jan

volume: 14

issue: 1

pages: e21574

abstract: INTRODUCTION: It has been observed that 5% of adolescents are affected by pubertal timing disorders. However, there is limited data about this in Pakistan. This cross-sectional study aimed to observe the patterns and causes of delayed puberty (DP) among patients presenting at the endocrine clinic of a tertiary care

hospital in Karachi. METHODS: This observational study was conducted at the endocrine clinic of Jinnah Postgraduate Medical Centre (JPMC) Unit II from 2007 to 2015. A detailed history was obtained from patients presenting with DP. We noted the available demographic data, main complaints, and family history of DP. Physical examinations were performed and the data recorded. Tanner staging was used to assess pubertal development. Relevant laboratory and imaging investigations were performed; data analysis was performed using SPSS 17 (IBM Corp., Armonk, NY). RESULTS: A total of 2670 patients were registered in the endocrine clinic during the study period, of which 171 presented with DP; 119 were males and 52 were females. There was a wide variation in age at presentation ranging from 10 to 32 years. The majority of patients presented with short stature - 69 (57.98%) males and 19 (36.53%) females. Small testes were present in 28 patients (23.52%); 19 (15.96%) males presented with absent secondary sexual characteristics and infertility was present in three (2.54%) males, primary amenorrhea was observed in 25 (48.07%), both primary amenorrhea and short stature were the presenting symptoms of five (9.61%), and failure of breast development was seen in three (5.76%) females. Constitutional delayed growth and puberty (CDGP) was diagnosed in 42 patients (24.6%). The definitive diagnosis of idiopathic hypogonadotropic hypogonadism (IHH) was made in 18 (10.5%) patients. In another 18 (10.5%) patients, we could not differentiate between CDGP and IHH. Functional hypogonadotropic hypogonadism (FHH) due to non-endocrine illness was present in 16 patients (9.4%). The cause of DP was hypogonadotropic hypogonadism in 33 (19.3%) patients whereas 44 patients presenting with DP could not be classified due to incomplete data. CONCLUSION: This study showed that CDGP was the most common cause of DP in our patients with the most common presentation being short stature in males and amenorrhea in females. It is essential to differentiate CDGP in children from a small fraction of the pathological and treatable causes of DP.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8866913/

Record 618

pmcid: PMC8777476

pmid: 34632855

doi: 10.1177/15353702211049361

title: Combining contrast-enhanced ultrasound and blood cell analysis to improve diagnostic accuracy of plasma cell mastitis.

authors

- Zheng Y
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journal: Experimental biology and medicine (Maywood, N.J.)

journalAbbr: Exp Biol Med (Maywood)

year: 2022

pubdate: 2022 Jan

volume: 247

issue: 2

pages: 97-105

abstract: Plasma cell mastitis is a benign suppurative disease of the breast, lack of specific clinical

manifestations, which is easy to be misdiagnosed and mistreated, often confused with mastitis, breast cancer (BC), and other diseases. Thus, we aimed to establish a combined model of promoting diagnostic accuracy of plasma cell mastitis by contrast-enhanced ultrasound (CEUS) patterns and routine blood cell analysis. Eighty-eight plasma cell mastitis, 91 breast cancer, and 152 other benign breast diseases' patients grouped according to pathological diagnosis underwent CEUS and blood cell analysis examination; 100 healthy female donors were involved. All the plasma cell mastitis and breast cancer patients presented hyperenhancement of CEUS breast lesions compared with others. The majority of plasma cell mastitis (65/88) showed perfusion defect of CEUS patterns with smooth edge (56/65) and multiple lesions (49/65); in contrast, fewer breast cancer patients (30/91) displayed perfusion defect. White blood cell count (WBC), neutrophils, and neutrophils/lymphocytes ratio of blood cell analysis in plasma cell mastitis patients increased significantly compared with other patients ($P < 0.0001$). Combining perfusion defect of CEUS patterns and WBC yielded an area under the receiver operating characteristic curve of 0.831, higher than single 0.720 and 0.774, respectively. The cut-off value of WBC ($7.28 \times 10\ 9$ /L) helped remaining 65.2% (15/23) atypical cases to be correctly diagnosed as plasma cell mastitis, not misdiagnosed as breast cancer. In conclusion, CEUS presented a clear perfusion defect pattern of plasma cell mastitis lesion for the first time. A precise WBC by routine blood cell analysis test can assist CEUS examination in the differential diagnosis of plasma cell mastitis and breast cancer. It is a promised combination for laboratory diagnostic of PCM.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8777476/

Record 619

pmcid: PMC9045542

pmid: 35832665

doi: 10.1055/s-0042-1744422

title: Comprehensive Evaluation of the Current Knowledge on Breast Implant Associated-Anaplastic Large Cell Lymphoma.

authors

- Yoo H
- Park JU
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journal: Archives of plastic surgery

journalAbbr: Arch Plast Surg

year: 2022

pubdate: 2022 Mar

volume: 49

issue: 2

pages: 141-149

abstract: Breast implant-associated anaplastic large cell lymphoma (BIA-ALCL) is a recently spotlighted T-cell origin non-Hodgkin's lymphoma with an increasing incidence of over 800 cases and 33 deaths reported worldwide. Development of BIA-ALCL is likely a complex process involving many factors, such as the textured implant surface, bacterial biofilm growth, immune response, and patient genetics. As the incidence of BIA-ALCL is expected to increase, it is important for all surgeons and physicians to be aware of this disease entity and acquire thorough knowledge of current evidence-based guidelines and recommendations. Early detection, accurate diagnosis, and appropriate treatment are the foundations of current care.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9045542/

Record 620

pmcid: PMC9948521

pmid: 35793692

doi: 10.1055/s-0042-1751063

title: Contraceptive Counseling for the Transgender Patient Assigned Female at Birth.

authors

- Okano SHP
- Pellicciotta GGM
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journal: Revista brasileira de ginecologia e obstetricia : revista da Federacao Brasileira das Sociedades de Ginecologia e Obstetricia

journalAbbr: Rev Bras Ginecol Obstet

year: 2022

pubdate: 2022 Sep

volume: 44

issue: 9

pages: 884-890

abstract: Although almost 0.7% of the Brazilian population identifies as transgender, there is currently no training for healthcare professionals to provide comprehensive care to these patients, including the discussion of reproductive planning. The use of testosterone promotes amenorrhea in the first months of use; however, this effect does not guarantee contraceptive efficacy, and, consequently, increases the risks of unplanned pregnancy. The present article is an integrative review with the objective of evaluating and organizing the approach of contraceptive counseling for the transgender population who were assigned female at birth. We used the PubMed and Embase databases for our search, as well as international guidelines on care for the transgender population. Of 88 articles, 7 were used to develop the contraceptive counseling model. The model follows the following steps: 1. Addressing the information related to the need for contraception; 2. Evaluation of contraindications to the use of contraceptive methods (hormonal and nonhormonal); and 3. Side effects and possible discomfort associated with the use of contraception. The contraceptive counseling model is composed of 18 questions that address the indications and contraindications to the use of these methods, and a flowchart to assist patients in choosing a method that suits their needs.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9948521/

Record 621

pmcid: PMC9154794

pmid: 35645173

doi: 10.1080/13880209.2022.2068617

title: Deer antler based active ingredients have protective effects on LPS/d-GalN-induced acute liver injury in mice through MAPK and NF-κB signalling pathways.

authors

- He G
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- Sun J

journal: Pharmaceutical biology

journalAbbr: Pharm Biol

year: 2022

pubdate: 2022 Dec

volume: 60

issue: 1

pages: 1077-1087

abstract: CONTEXT: Deer antler based active ingredients are known to have certain anti-inflammatory and antioxidant activities. However, its potential hepatoprotective effect remains unclear. OBJECTIVE: This article reports the hepatoprotective effect of protein components in deer antler bases (R1) on lipopolysaccharide/d-galactosamine (LPS/d-GalN)-induced acute liver injury (ALI) in mice, and explores its possible mechanism. MATERIALS AND METHODS: The four separated and purified protein components of deer antler bases were screened and verified by the RAW264.7 cell inflammation model. In the in vivo experiment of LPS/d-GalN-induced ALI in mice, ALT, AST, SOD, CAT, GSH and MDA were detected. The liver histopathology was analysed, the COX-2 and iNOS proteins were analysed by immunohistochemistry, and 4-HNE was analysed by immunofluorescence staining. In addition, the effects on the MAPK pathway and NF-κB/IκB-α pathway in liver proteins were explored. RESULTS: With isolated RA protein fraction pre-treated RAW264.7 cells, NO production decreased by 35.3% compared with the model group. The experimental results of ALI in mice induced by LPS/d-GalN show that R1 protein components can protect mice from ALI through anti-inflammatory and anti-oxidative stress effects and reduce liver pathological damage in mice. The results also indicate that the R1 protein component may protect the liver by inhibiting the activation of the MAPK pathway and the NF-κB/IκB-α pathway induced by LPS/d-GalN. CONCLUSIONS: The separated and purified R1 protein component of deer antler base has a good protective effect on LPS/d-GalN-induced liver injury, and may become a potential material for protecting against liver injury.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9154794/

Record 622

pmcid: PMC9714018

pmid: 36451171

doi: 10.1186/s13006-022-00523-3

title: Determinants of delayed onset of lactogenesis II among women who delivered via Cesarean section at a tertiary hospital in China: a prospective cohort study.

authors

- Lian W
- Ding J
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journal: International breastfeeding journal

journalAbbr: Int Breastfeed J

year: 2022

pubdate: 2022 Nov 30

volume: 17

issue: 1

pages: 81

abstract: BACKGROUND: Cesarean birth is associated with a higher prevalence of delayed onset of lactogenesis II (DOLII) than vaginal birth. DOLII refers to the delayed initiation of copious milk production beyond 72 h after birth. This study aimed to determine the prevalence of, and factors associated with, DOLII among women who delivered via Cesarean section in China. METHODS: This prospective longitudinal cohort study recruited 468 women who delivered via Cesarean section at a tertiary hospital in China from 9 October 2021 to 17 May 2022. Face-to-face interviews were conducted during their delivery hospital stay to obtain information about demographic, medical, and breastfeeding factors. We assessed the onset of lactogenesis on postpartum day four, based on the maternal perception of changes in breast fullness. The Edinburgh Postnatal Depression Scale (EPDS) was used to screen for postpartum depression. Women with DOLII were interviewed via telephone or WeChat daily for one week postpartum to determine the timing of the onset of lactogenesis II. Univariate and multivariable logistic regression analyses were used to identify the determinants of DOLII. RESULTS: DOLII was experienced by 156 of 468 participants (33.3%). After adjusting for potential confounders, the odds of DOLII were 95% higher in primiparous women than multiparous women (adjusted odds ratio [aOR] 1.95; 95% confidence interval [CI] 1.29, 2.98), 75% higher in women with a serum albumin concentration < 35 g / L than women with normal serum albumin concentrations (aOR 1.78; 95% CI 1.09, 2.99), increased by 2.03-fold in women with an EPDS score ≥ 10 than women with an EPDS score < 10 (aOR 2.03; 95% CI 1.35, 3.07), and decreased in women with a higher number of breastfeeding sessions in the first 48 h postpartum (aOR 0.88; 95% CI 0.83, 0.93). CONCLUSIONS: One-third of women with Cesarean section delivery experienced DOLII. DOLII was more likely in women who were primiparous, had a serum albumin concentration < 35 g / L, had a lower frequency of breastfeeding sessions, and had an EPDS score ≥ 10. Women with these risk factors who deliver via Cesarean section may need early breastfeeding support to ensure successful lactation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9714018/

Record 623

pmcid: PMC8888017

pmid: 35252796

doi: 10.1002/iju5.12409

title: Development of male breast cancer in a patient with prostate cancer during androgen deprivation therapy.

authors

- Mizusawa H
- Komatsu A
- Mimura Y
- Maejima T

journal: IJU case reports

journalAbbr: IJU Case Rep

year: 2022

pubdate: 2022 Mar

volume: 5

issue: 2

pages: 118-121

abstract: INTRODUCTION: Male breast cancer accounts for <1% of all breast cancer. We report a patient with prostate cancer during hormone therapy who developed breast cancer. CASE PRESENTATION: An 88-year-old male underwent androgen deprivation therapy for prostate cancer and developed an induration in the left breast 7 years after the start of treatment. After close examination, he was diagnosed with left breast cancer with lymph node metastasis. The prostate cancer was stable in a hormone-sensitive state. Left mastectomy was performed and the pathological diagnosis was invasive ductal carcinoma. DISCUSSION: In addition to our patient, seven patients who developed breast cancer during hormone therapy for prostate cancer were examined. Five of six

patients had stage II or above, and four patients had lymph node metastases. Although local breast symptoms are frequently observed as adverse effects of hormone therapy, caution is warranted regarding male breast cancer.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8888017/

Record 624

pmcid: PMC9473749

pmid: 36120247

doi: 10.7759/cureus.28041

title: Diagnosis of Intrauterine Pregnancy Despite the Presence of Etonogestrel Implant.

authors

- Sawyers RJ
- Parikh PM
- Lazaro JJ
- Greenberg MR

journal: Cureus

journalAbbr: Cureus

year: 2022

pubdate: 2022 Aug

volume: 14

issue: 8

pages: e28041

abstract: While contraception is an important method to avoid pregnancy, it is not always effective. Our case details a 33-year-old-female with an etonogestrel implant who presented to the emergency department (ED) with a two-week history of vomiting and abdominal pain. Pelvic and transvaginal ultrasound confirmed a single, live intrauterine pregnancy. Our case serves as a reminder that ED providers should have a high index of suspicion for pregnancy in clinically relevant scenarios, despite contraceptive methods, until the appropriate confirmatory diagnostic evaluation for pregnancy is completed.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9473749/

Record 625

pmcid: PMC8727246

pmid: 35071540

doi: 10.12998/wjcc.v10.i1.361

title: Diagnostic and surgical challenges of progressive neck and upper back painless masses in Madelung's disease: A case report and review of literature.

authors

- Yan YJ
- Zhou SQ
- Li CQ
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journal: World journal of clinical cases

journalAbbr: World J Clin Cases

year: 2022

pubdate: 2022 Jan 7

volume: 10

issue: 1

pages: 361-370

abstract: BACKGROUND: Madelung's disease (MD) is a chronic alcoholism-associated metabolic syndrome characterized by symmetrical subcutaneous deposition of adipose tissue in the head, neck, shoulders, back, trunk, and nerve roots of the upper and lower limbs. It is relatively rare in Asian individuals and is prone to misdiagnosis. Herein, we report a case of a patient with MD who had undergone surgical management at our hospital, and we discuss the pathogenesis, diagnosis, and treatment of MD. CASE SUMMARY: We report a case of MD in a 65-year-old man of Han descent. The patient had multiple, painless progressive masses for more than five years in the neck and more than 30 years in the upper back. Because of neck mobility limitations and progressive cosmetic deformities caused by the masses, he was admitted to our hospital. He drank approximately 500 mL of liquor per day and smoked heavily for more than 30 years. Contrast-enhanced computed tomography of the neck and chest documented abundant unencapsulated, subcutaneous fatty deposits. We prepared a staged operation plan. The patient was diagnosed with MD; he was advised to abstain from alcohol and was followed up regularly. After a 3-month follow-up, no recurrence of fat accumulation was found in the surgical areas. CONCLUSION: This report presents a case of surgical treatment for MD to improve clinicians' understanding of the disease.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8727246/

Record 626

pmcid: PMC9031563

pmid: 35245508

doi: 10.1016/S2352-3018(21)00326-X

title: Disparities in breast cancer survival between women with and without HIV across sub-Saharan Africa (ABC-DO): a prospective, cohort study.

authors

- Chasimpha S
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journal: The lancet. HIV

journalAbbr: Lancet HIV

year: 2022

pubdate: 2022 Mar

volume: 9

issue: 3

pages: e160-e171

abstract: BACKGROUND: Studies have shown increased mortality among women living with HIV diagnosed with breast cancer compared with HIV-negative women with breast cancer. We aimed to examine how this HIV differential varies by patient or breast tumour characteristics. METHODS: The African Breast Cancer-Disparities in Outcomes (ABC-DO) study is a prospective cohort of women (aged ≥18 years) with incident breast cancer recruited consecutively at diagnosis (2014-17) from hospitals in Namibia, Nigeria, South Africa, Uganda, and Zambia. Detailed clinical and epidemiological data, including self-reported or tested HIV status, were collected at baseline. Participants were actively followed up via telephone calls every 3 months. The primary outcome was all-cause mortality, assessed in all women who had at least one updated vital status after baseline interview. Using Cox regression, we examined differences in overall survival by HIV status in the cohort, and across country and patient subgroups, adjusted for age, tumour grade, and tumour stage at cancer diagnosis. FINDINGS: Between Sept 8, 2014, and Dec 31, 2017, we recruited 2154 women with primary breast cancer, 519 of whom were excluded due to their countries having small numbers of women with HIV for comparison. Among the remaining 1635 women, 313 (19%) were living with HIV, 1184 (72%) were HIV negative, and 138 (9%) had unknown HIV status. At breast cancer diagnosis, women with HIV were younger and had lower body-mass index (BMI) than their HIV-negative counterparts, but had similar tumour stage, grade, and receptor subtypes. At the end of the follow-up (Jan 1, 2019), a higher proportion of women with HIV (137 [44%] of 313) had died than had HIV-negative women (432 [37%] of 1184). Crude 3-year survival was 9% lower for women with HIV (46% [95% CI 40-53]) than for HIV-negative women (55% [52-59]; hazard ratio (HR) 1·41 [1·15-1·74]). The HIV survival differential did not differ by age, BMI, tumour subtype, or tumour grade, but was stronger in women with non-metastatic disease (3-year survival 52% HIV-positive vs 63% HIV-negative women, adjusted HR 1·65 [1·30-2·10]), whereas women with metastatic cancer had low survival, regardless of HIV status. INTERPRETATION: The larger survival deficit among women with HIV with non-metastatic breast cancer calls for a better understanding of the reasons underlying this differential (eg, biological mechanisms, health behaviours, detrimental HIV-breast cancer treatment interactions, or higher HIV background mortality) to inform strategies for reducing mortality among this patient group. FUNDING: Susan G Komen, International Agency for Research on Cancer, National Cancer Institute, and UK-Commonwealth Scholarships.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9031563/

Record 627

pmcid: PMC9763905

pmid: 36478423

doi: 10.5021/ad.21.180

title: Efficacy and Safety of Cold-Induced Noninvasive Targeted Fat Reduction in Pseudogynecomastia.

authors

- Hong JY
- Park SJ
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journal: Annals of dermatology

journalAbbr: Ann Dermatol

year: 2022

pubdate: 2022 Dec

volume: 34

issue: 6

pages: 412-418

abstract: BACKGROUND: Treatment options for pseudogynecomastia are limited, and the demand for noninvasive breast fat reduction is increasing. OBJECTIVE: We evaluated the efficacy and safety of a cold-induced lipolysis device for treating pseudogynecomastia. METHODS: In this 16-week prospective trial, a total of 15 male patients with pseudogynecomastia were treated twice with cryolipolysis. The primary endpoint was a change in the chest circumference from baseline at posttreatment week 8. Secondary endpoints were changes in body weight, fat thickness assessed using ultrasonography, independent evaluator- and patient-rated improvement, and Simon's gynecomastia class (SGC) grading. RESULTS: The primary assessment, a reduction of 3.05 cm in the mean chest circumference at 8 weeks post-treatment compared to baseline, was statistically significant. The treatment effect was cumulative, with a steady decrease in chest circumference and fat thickness over the 16-week study period. The mean pain score immediately after the first session of treatment was 2.0±1.36, based on a scale of 0~10, with a score of 10 being the worst pain ever experienced. The pain decreased substantially after the end of the procedure. CONCLUSION: Cryolipolysis was demonstrated to be an effective and safe option for reducing breast fat in pseudogynecomastia. Male with mild to moderate breast enlargement without skin excess can be ideal candidates.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9763905/

Record 628

pmcid: PMC10426231

pmid: 35869406

doi: 10.1007/s42000-022-00388-5

title: Endocrine disruptors in ancient times and Greek mythology: the spring of Salmacis and the curse of Hermaphroditus.

authors

- Vasileiou V
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- Georgopoulos NA

journal: Hormones (Athens, Greece)

journalAbbr: Hormones (Athens)

year: 2022

pubdate: 2022 Dec

volume: 21

issue: 4

pages: 729-734

abstract: According to Greek mythology, the spring waters of Salmacis (or Salmakis) feminized the god Hermaphroditus (or Hermaphroditos) and transformed his nature from male to half-male and half-female. The mythical properties of these waters are described in the writings of authors and philosophers of the Hellenistic period. It is evident that the spring of Salmacis and lake actually existed (located in Halicarnassus, today Bodrum, Turkey) and are not the product of poetic imagination. Hence, it could be hypothesized that there were certain natural elements in the waters that had a feminizing effect on the male reproductive axis. We now know, in fact, that naturally occurring environmental agents, also known as endocrine disruptors, can affect the endocrine and reproductive function of both males and females. However, since most endocrine disruptors

today are manmade products of the modern industrial lifestyle, the presence and effect of naturally occurring disruptors in times preceding the Industrial Revolution are not widely discussed. It is thus against this background that we seek to formulate a differential diagnosis of male feminization attributable to the effect of natural environmental factors in the form of endocrine disruptors that will have existed in environments round the globe since time immemorial. We conclude that if there had been an accumulation of the mycotoxin zearalenone (ZEA) in the waters of Salmacis, chronic exposure to the lake's water could have resulted in the phenotypic changes described in the Salmacis myth.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10426231/

Record 629

pmcid: PMC9204619

pmid: 35708696

doi: 10.1001/jamaoncol.2022.1641

title: Enzalutamide Monotherapy vs Active Surveillance in Patients With Low-risk or Intermediate-risk Localized Prostate Cancer: The ENACT Randomized Clinical Trial.

authors

- Shore ND
- Renzulli J
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- Silberstein J
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- Tomlins SA

journal: JAMA oncology

journalAbbr: JAMA Oncol

year: 2022

pubdate: 2022 Aug 1

volume: 8

issue: 8

pages: 1128-1136

abstract: IMPORTANCE: There are few published studies prospectively assessing pharmacological interventions that may delay prostate cancer progression in patients undergoing active surveillance (AS). OBJECTIVE: To compare the efficacy and safety of enzalutamide monotherapy plus AS vs AS alone in patients with low-risk or intermediate-risk prostate cancer. DESIGN, SETTING, AND PARTICIPANTS: The ENACT study was a phase 2, open-label, randomized clinical trial conducted from June 2016 to August 2020 at 66 US and Canadian sites. Eligible patients were 18 years or older, had received a diagnosis of histologically proven low-risk or intermediate-risk localized prostate cancer within 6 months of screening, and were undergoing AS. Patients were monitored during 1 year of treatment and up to 2 years of follow-up. Data analysis was conducted in February 2021. INTERVENTIONS: Randomized 1:1 to enzalutamide, 160 mg, monotherapy for 1 year or continued AS, as stratified by cancer risk and follow-up biopsy type. MAIN OUTCOMES AND MEASURES: The primary end point was time to pathological or therapeutic prostate cancer progression (pathological, ≥1 increase

in primary or secondary Gleason pattern or ≥15% increased cancer-positive cores; therapeutic, earliest occurrence of primary therapy for prostate cancer). Secondary end points included incidence of a negative biopsy result, percentage of cancer-positive cores, and incidence of a secondary rise in serum prostate-specific antigen (PSA) levels at 1 and 2 years, as well as time to PSA progression. Adverse events were monitored to assess safety. RESULTS: A total of 114 patients were randomized to treatment with enzalutamide plus AS and 113 to AS alone; baseline characteristics were similar between treatment arms (mean [SD] age, 66.1 [7.8] years; 1 Asian individual [0.4%], 21 Black or African American individuals [9.3%], 1 Hispanic individual [0.4%], and 204 White individuals [89.9%]). Enzalutamide significantly reduced the risk of prostate cancer progression by 46% vs AS (hazard ratio, 0.54; 95% CI, 0.33-0.89; P = .02). Compared with AS, odds of a negative biopsy result were 3.5 times higher; there was a significant reduction in the percentage of cancer-positive cores and the odds of a secondary rise in serum PSA levels at 1 year with treatment with enzalutamide; no significant difference was observed at 2 years. Treatment with enzalutamide also significantly delayed PSA progression by 6 months vs AS (hazard ratio, 0.71; 95% CI, 0.53-0.97; P = .03). The most commonly reported adverse events during enzalutamide treatment were fatigue (62 [55.4%]) and gynecomastia (41 [36.6%]). Three patients in the enzalutamide arm died; none were receiving the study drug at the time of death. No deaths were considered treatment-related. CONCLUSIONS AND RELEVANCE: The results of this randomized clinical trial suggest that enzalutamide monotherapy was well-tolerated and demonstrated a significant treatment response in patients with low-risk or intermediate-risk localized prostate cancer. Enzalutamide may provide an alternative treatment option for patients undergoing AS. TRIAL REGISTRATION: ClinicalTrials.gov Identifier: NCT02799745.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9204619/

Record 630

pmcid: PMC9859683

pmid: 36683888

doi: 10.1055/s-0042-1759498

title: Etiopathological Factors Associated with Gynecomastia Patients Seeking Surgical Correction in the South Indian Population.

authors

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- Rajan S
- Anu AK

journal: Indian journal of plastic surgery : official publication of the Association of Plastic Surgeons of India

journalAbbr: Indian J Plast Surg

year: 2022

pubdate: 2022 Dec

volume: 55

issue: 4

pages: 364-367

abstract: Background Although several medical conditions are associated with gynecomastia, around 60% of cases are idiopathic. The role of endocrine testing in idiopathic gynecomastia is controversial. This study was done to determine the etiological and lifestyle factors associated with different grades of gynecomastia. Patients and Methods This was a descriptive study conducted at the department of plastic surgery in a tertiary care hospital in South India between January 2014 and December 2016, among the patients seeking corrective surgery for gynecomastia. Results A total of 73 patients were included in the study with a mean age of 22.56 years. The majority of the patients displayed Simon's Grade IIa gynecomastia (56.2%). The etiological factors identified in this study were hormonal abnormalities (47.95%), hypogonadism (2.7%), and drug intake (1.4%). The most common hormonal abnormality discovered was high estradiol values (15.1%) followed by decreased testosterone levels (13.7%). The chi-squared test revealed no statistically significant relationship between the hormonal values, type of food, body mass index (BMI), or physical activity and the grades of gynecomastia.

Conclusion Most of the patients (50.68%) in our study had idiopathic gynecomastia. The most common hormonal abnormality detected was high estradiol values. No correlation was found between the hormonal values, type of food, BMI, or physical activity and the grades of gynecomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9859683/

Record 631

pmcid: PMC9692590

pmid: 36422956

doi: 10.3390/toxins14110782

title: Evaluation of Zearalenones and Their Metabolites in Chicken, Pig and Lamb Liver Samples.

authors

- Llorens Castelló P
- Sacco MA
- Aquila I
- Moltó Cortés JC
- Juan García C

journal: Toxins

journalAbbr: Toxins (Basel)

year: 2022

pubdate: 2022 Nov 11

volume: 14

issue: 11

pages:

abstract: Zearalenone (ZON), zearalanone (ZAN) and their phase I metabolites: α-zearalenol (α-ZOL), β-zearalenol (β-ZOL), α-zearalalanol (α-ZAL) and β-zearalalanol (β-ZAL) are compounds with estrogenic activity that are metabolized and distributed by the circulatory system in animals and can access the food chain through meat products from livestock. Furthermore, biomonitoring of zearalenones in biological matrices can provide useful information to directly assess mycotoxin exposure; therefore, their metabolites may be suitable biomarkers. The aim of this study was to determine the presence of ZON, ZAN and their metabolites in alternative biological matrices, such as liver, from three different animals: chicken, pig and lamb, in order to evaluate their exposure. A solid-liquid extraction procedure coupled to a GC-MS/MS analysis was performed. The results showed that 69% of the samples were contaminated with at least one mycotoxin or metabolite at varying levels. The highest value (max. 152.62 ng/g of β-ZOL) observed, and the most contaminated livers (42%), were the chicken liver samples. However, pig liver samples presented a high incidence of ZAN (33%) and lamb liver samples presented a high incidence of α-ZOL (40%). The values indicate that there is exposure to these mycotoxins and, although the values are low (ranged to 0.11-152.6 ng/g for α-ZOL and β-ZOL, respectively), analysis and continuous monitoring are necessary to avoid exceeding the regulatory limits and to control the presence of these mycotoxins in order to protect animal and human health.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9692590/

Record 632

pmcid: PMC8977476

pmid: 35386270

doi: 10.2147/RRU.S303215

title: Evolution of Androgen Deprivation Therapy (ADT) and Its New Emerging Modalities in Prostate Cancer: An Update for Practicing Urologists, Clinicians and Medical Providers.

authors

- Choi E
- Buie J
- Camacho J
- Sharma P
- de Riese WTW

journal: Research and reports in urology

journalAbbr: Res Rep Urol

year: 2022

pubdate: 2022

volume: 14

issue:

pages: 87-108

abstract: Androgen deprivation therapy (ADT) has been the main management strategy for prostate cancer for more than eight decades, nowadays achieved commonly by administration of luteinizing hormone-releasing hormone agonists. ADT markedly suppresses androgen hormones with the long-term risks of adverse events such as muscle weakness, impairment of glucose and lipid metabolism, impotence, osteoporosis, and secondary fractures. Extensive research has provided significantly better insight into the dynamics of ADT including identification of the benefits of sequential and combination therapies. This has led to the development of new pharmaceutical ADT modalities. This review provides a general overview of the evolution of ADT in the context of the new emerging pharmaceutical ADT modalities so that clinicians and medical providers have a better understanding of personalizing the available ADT options with their different risk-benefit profiles.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8977476/

Record 633

pmcid: PMC8775379

pmid: 35052243

doi: 10.3390/healthcare10010079

title: Experience of Late Miscarriage and Practical Implications for Post-Natal Health Care: Qualitative Study.

authors

- Kukulskienė M
- Žemaitienė N

journal: Healthcare (Basel, Switzerland)

journalAbbr: Healthcare (Basel)

year: 2022

pubdate: 2022 Jan 1

volume: 10

issue: 1

pages:

abstract: Miscarriage is the most common reason for pregnancy loss, affecting around one in four pregnancies. It is classified as a traumatic event, associated with an increased risk for depression, anxiety, post-traumatic stress, alcohol dependence, somatic symptoms, sexual dysfunction, suicide, and complicated grief. This study aimed to analyse experiences of late miscarriage and to describe practical implications for post-natal health care based on characteristics of pregnancy loss revealed in a qualitative study. Seven women who had late

miscarriages participated in in-depth biographical interviews. A phenomenological thematic analysis was applied. Experiential characteristics of late miscarriage were described by four themes and 13 subthemes: the initial splitting state (Dissociation, An Opened Void, An impaired Symbiosis, and The Body is Still Pregnant while the Psyche is Mourning); Betrayal of the body (Symbolic Experience of Internalised Death, Shocking Materiality of the Ongoing Miscarriage, Lost control of the Body, and Confusing Body Signals); Disconnecting (Depersonalising Medical Environment, Guilt Falsifies perception, and Retreat as a means of Self-Preservation); and Reconnecting (Collecting Shatters and Reinterpretation of Maternal Identity). Based on the results of the experiential analysis, another four themes represent practical recommendations for post-natal health care: Informing, Opportunity for Goodbye, Attention to Emotional Wellbeing, and Respectful Hospital Environment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8775379/

Record 634

pmcid: PMC9705658

pmid: 35106735

doi: 10.1007/s40477-022-00663-6

title: Experimental evaluation of high intensity focused ultrasound for fat reduction of ex vivo porcine adipose tissue.

authors

- Filippou A
- Damianou C

journal: Journal of ultrasound

journalAbbr: J Ultrasound

year: 2022

pubdate: 2022 Dec

volume: 25

issue: 4

pages: 815-825

abstract: PURPOSE: The present study was stimulated by the continuous growth of commercially available high intensity focused ultrasound (HIFU) systems for fat reduction. Herein, HIFU was utilised for fat ablation using a single-element ultrasonic transducer operating in thermal mode. METHODS: The custom-made concave transducer that operates at 1.1 MHz was assessed on excised porcine adipose tissue for its ability to reduce fat. Ultrasonic sonications were executed on the adipose tissue utilising acoustical power between 14 and 75 W and sonication time in the range of 1-10 min. The mass of the adipose tissue sample was weighed afore and after ultrasonic sonications and the effect of the sonication on the mass change was recorded. RESULTS: Mass change was linearly dependent with either increasing acoustical power or sonication time and was in the range of 0.46-1.9 g. High acoustical power of 62.5 W for a sonication time of 1 min and a power of 75 W for a sonication time of 5 min, respectively resulted in the formation of a lesion or possible cavitation on the piece of excised adipose tissue. CONCLUSION: The study demonstrated the efficacy of the proposed transducer in achieving a reduction of excised fat tissue. The present findings indicate the potential use of the transducer in a HIFU system indicated for the reduction of subcutaneous adipose tissue where increased values of acoustical power can result in increased amounts of fat reduction.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9705658/

Record 635

pmcid: PMC8900075

pmid: 34380293

doi: 10.4274/jcrpe.galenos.2021.2021.0170

title: Feminizing Adrenocortical Tumors as a Rare Etiology of Isosexual/Contrasexual Pseudopuberty.

authors

- Vurallı D
- Gönç N
- Özön A
- Ekinci S
- Doğan HS
- Tekgül S
- Alikaşifoğlu A

journal: Journal of clinical research in pediatric endocrinology

journalAbbr: J Clin Res Pediatr Endocrinol

year: 2022

pubdate: 2022 Mar 3

volume: 14

issue: 1

pages: 17-28

abstract: OBJECTIVE: Estrogen-secreting adrenocortical tumors (ACTs) are quite rare with feminizing adrenocortical tumors (FATs) accounting for 0.37-2% of all ACTs. The aim was to evaluate clinical and hormonal characteristics of FATS as well as treatment options and follow-up in the pediatric age group. METHODS: Medical records of children with ACTs presenting to a single center in the last two decades were reviewed. Literature review within Pubmed revealed 34 pediatric patients (22 boys) with FAT among 192 articles. RESULTS: Among the 25 children presenting with ACTs in the last two decades, two new pediatric cases of FAT were identified, one benign and the other malignant, in two genders with different clinical presentations. Literature review showed that FATs are extremely rare tumors that are most commonly seen in men and boys presenting with gynecomastia. FATs are more common in children ≤8 years of age, with a median age at diagnosis of six years. While boys present with contrasexual pseudopuberty signs, girls present with isosexual pseudopuberty. A high estrogen level strongly supports diagnosis, while elevations in other adrenal hormones may be seen. FATs are usually malignant in adults and prognosis is generally very poor. However, in children approximately half are benign although assessment of malignant potential depends on clinical behavior of the tumor. FATs are very unpredictable so even after surgery long-term follow-up is required. FATs presenting in childhood may have a better prognosis than adult presentation tumors as most FATs in children are followed without recurrence of tumor. CONCLUSION: FATs are more common in children ≤8 years of age, with a median age at diagnosis of six years. FATs in childhood may have a better prognosis than in adult males.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8900075/

Record 636

pmcid: PMC8837037

pmid: 35160095

doi: 10.3390/jcm11030642

title: Genetic Factors of Idiopathic Gigantomastia: Clinical Implications of Aromatase and Progesterone Receptor Polymorphisms.

authors

- Kasielska-Trojan A
- Pietrusiński M

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- Strużyna J
- Borowiec M
- Antoszewski B

journal: Journal of clinical medicine

journalAbbr: J Clin Med

year: 2022

pubdate: 2022 Jan 27

volume: 11

issue: 3

pages:

abstract: The role of estrogen, progesterone, their receptors and aromatase in the development of the breast is well documented. In this study we examined the association of genetic variants of progesterone receptor (PGR) and aromatase (CYP19A1) genes with gigantomastia risk. We conducted a case-control study among 124 women: 60 with gigantomastia and 64 controls. We examined the single nucleotide polymorphisms (SNPs) for CYP19A1 (rs749292 and rs7172156) and PGR (rs1042838). Our results showed that allele G in rs749292 (CYP19A1) increased the risk of gigantomastia, but not significantly (p = 0.09). There is a correlation between rs1042838 (PGR) and waist-to-hip ratio (WHR) in women with gigantomastia-AC genotype correlates with lower WHR and CC with higher WHR. There were no correlations between the onset of gigantomastia, the age of menarche and the length of the menstrual cycle, and rs1042838, rs749292 and rs7172156. We did not find differences in the SNP of PGR (rs1042838) between women with gigantomastia and controls. However, our findings showed more frequent G allele in CYP19A1 (rs749292) in women with gigantomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8837037/

Record 637

pmcid: PMC9722749

pmid: 36483467

doi: 10.3389/fped.2022.1042427

title: Gonadal Y-chromosome mosaicism with 45, X Turner syndrome complicated with bilateral HCG-secreting gonadoblastoma.

authors

- Zheng R
- Chen Q
- Ma H
- Liu J
- Chen H
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- Chen H
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- Li Y
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journal: Frontiers in pediatrics

journalAbbr: Front Pediatr

year: 2022

pubdate: 2022

volume: 10

issue:

pages: 1042427

abstract: We report a rare case of bilateral HCG-secreting gonadoblastomas (Gb) in a 5.25-year-old girl of 45, X Turner syndrome (TS) with gonadal Y chromosome mosaicism. The clinical data were summarized, and the literatures were reviewed. The patient had enlarged breasts for 2 years and 3 months, with elevated β-HCG of blood found for 8 months. The level of β-HCG of cerebrospinal fluid, cranial MRI, chest and abdominal CT, and pelvic MRI were normal. After surgical gonad exploration, biopsy and excision, gonad venous blood hormone examination and SRY gene detection of gonad tissue, the diagnosis was confirmed as HCG-secreting Gb (bilateral) and TS (45, X) with gonad Y chromosome mosaicism. The patient received 4 courses of chemotherapy, and regular outpatient follow-up. At 9 months after gonadectomy, there was no clinical, laboratory, or radiological evidence of recurrence. We reported a nonclassical case of 45, X Turner syndrome (TS) with gonadal Y chromosome mosaicism, who presented with breast development as the first manifestation and then virilization due to bilateral HCG-secreting gonadoblastomas. Detection of serum β-HCG and AFP is requisite for the diagnosis of precocious puberty, karyotyping is important for virilizing phenotypic female, and virilization in Turner syndrome implies the existence of Y chromosome(substance) (peripheral blood or tissue mosaicism) and the occurrence of gonadal tumors.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9722749/

Record 638

pmcid: PMC9800095

pmid: 36590381

doi: 10.1155/2022/6641661

title: Graves' Disease Presenting as a Unilateral Breast Mass.

authors

- Akhavan NN
- Maska E

journal: Case reports in medicine

journalAbbr: Case Rep Med

year: 2022

pubdate: 2022

volume: 2022

issue:

pages: 6641661

abstract: INTRODUCTION: Graves' disease is an autoimmune thyroid disorder that is the most common cause of hyperthyroidism. Common manifestations of Graves' disease include weight loss, palpitations, heat intolerance, fatigue, tremors, and exophthalmos, occurring in more than 50% of patients. In rare cases, findings may predominate in one organ system; isolated findings of diarrhea, anxiety, or gynecomastia (as in our case) may occur, distracting from the correct diagnosis. Case Report . We report on a 37-year-old male who presented to the primary care clinic with the chief complaint of a tender right-sided breast mass and with an associated loss of appetite and forty-pound weight loss. Laboratory evaluation revealed suppressed TSH and elevated free T4. A bilateral diagnostic mammogram revealed bilateral gynecomastia. A nuclear medicine thyroid uptake scan was subsequently ordered, which showed the diffusely enlarged thyroid gland with

homogenous increased uptake throughout, consistent with Graves' disease. CONCLUSION: It is important to keep a high index of suspicion for thyroid disease as hyperthyroid states may be deceiving in presenting with single organ system involvement. Although it is rare, gynecomastia is a known finding in Graves' disease and can be the presenting sign. Patients with unexplained gynecomastia or breast masses should be screened for thyrotoxicosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9800095/

Record 639

pmcid: PMC9647523

pmid: 36335424

doi: 10.12659/AJCR.937370

title: Gynecomastia and Malignancy: A Case of Male Invasive Ductal Breast Carcinoma Treated with Neoadjuvant Chemotherapy.

authors

- Mekheal E
- Kania BE
- Kumari P
- Kumar V
- Maroules M

journal: The American journal of case reports

journalAbbr: Am J Case Rep

year: 2022

pubdate: 2022 Nov 6

volume: 23

issue:

pages: e937370

abstract: BACKGROUND Male breast cancer represents a rare malignancy with identifiable risk factors, including genetics, radiation exposure, liver dysfunction, and concomitant diagnosis of Klinefelter syndrome. Gynecomastia can commonly present in these patients, and despite increased estrogen levels in adipose breast tissue, gynecomastia has not been proven to be a significant risk factor for carcinoma development. Male patients with new-onset breast masses are recommended to undergo diagnostic mammograms and breast ultrasound for further evaluation. Those diagnosed with breast cancer most commonly have invasive ductal carcinoma of the breast, and over half of these patients are found to have estrogen and progesterone receptor (ER/PR) positivity. CASE REPORT In this case report, we present a Black man with gynecomastia and an areolar lesion for a 6-month duration following a traumatic event. He was initially referred to the surgical team for further evaluation, and subsequent imaging and biopsy data revealed ER/PR-positive invasive ductal carcinoma. Multidisciplinary discussions were held, and the patient was arranged to begin neoadjuvant treatment with doxorubicin hydrochloride and cyclophosphamide, followed by treatment with paclitaxel (AC-T) chemotherapy, followed by bilateral mastectomy and adjuvant hormonal therapy. CONCLUSIONS The treatment of male breast cancer has remained relatively like that of female breast cancer, which may be due to the limited data in the treatment of male breast cancer. Thus far, studies involving neoadjuvant chemotherapy of female patients have demonstrated promising responses to expand surgical options for patients and possibly decrease the rates of recurrence. Additional studies are warranted to discern optimal therapy for the male patient population.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9647523/

Record 640

pmcid: PMC8811439

pmid: 35098766

doi: 10.1177/03000605221075815

title: Gynecomastia with rare granulomatous lobular mastitis: a case report and literature review.

authors

- Yin L
- Agyekum EA
- Zhang Q
- Wu T
- Qian X

journal: The Journal of international medical research

journalAbbr: J Int Med Res

year: 2022

pubdate: 2022 Jan

volume: 50

issue: 1

pages: 3000605221075815

abstract: This study was performed to describe a rare case of granulomatous lobular mastitis (GLM) that was successfully treated with bromocriptine in a male patient with gynecomastia and hyperprolactinemia. A 20-year-old man presented with a 1-year history of breast enlargement and galactorrhea. Physical examination revealed bilateral breast enlargement, porous discharge, and a 3-cm left breast lump in the 10-o'clock quadrant. Magnetic resonance imaging of the brain showed a 1.2-mm pituitary tumor. Laboratory analysis revealed hyperprolactinemia with low serum testosterone and elevated prolactin and estradiol levels. The lump in the left breast was examined by ultrasonography and mammography, and a core needle biopsy revealed chronic inflammation. The patient's galactorrhea and breast lump disappeared after 3 months of treatment with bromocriptine at 2.5 mg once a day. His serum prolactin level also normalized. Following a review of this case, the patient was diagnosed with gynecomastia with hyperprolactinemia complicated by rare GLM. To the best of our knowledge, this is the first reported case of concurrent gynecomastia and GLM.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8811439/

Record 641

pmcid: PMC9663914

pmid: 36389365

doi: 10.3389/fped.2022.978311

title: Gynecomastia: A systematic review of pharmacological treatments.

authors

- Berger O
- Landau Z
- Talisman R

journal: Frontiers in pediatrics

journalAbbr: Front Pediatr

year: 2022

pubdate: 2022

volume: 10

issue:

pages: 978311

abstract: BACKGROUND: Pubertal gynecomastia (PG), a benign condition with varied reported prevalence, typically appears at 13-14 years-old and is mostly idiopathic and self-limited. Psychologic impairments are common among adolescents with gynecomastia. Surgical intervention is reserved to severe cases and is offered towards the end of puberty. Pharmacological treatment is seldom given by clinicians mainly due to insufficient published data. We conducted this systematic literature review to assess the efficacy, safety, side effects, and complications of pharmacological treatments published. METHODS: MEDLINE, Embase, and Cochrane CENTRAL were searched for the terms "gynecomastia", "pubertal", and "adolescent" in conjunction with medications from the Selective Estrogen Receptor Modulator (SERM), aromatase inhibitors (AI), and androgens groups in different combinations to optimize the search results. Exclusion criteria included: studies based on expert opinion, similar evidence-based medicine levels studies, and studies which discuss gynecomastia in adults. Selected articles were assessed by two authors. Data collected included: the level of evidence, population size, treatment regimen, follow-up, outcomes, complications, and side effects. RESULTS: Of 1,425 published studies found and examined meticulously by the authors, only 24 publications met all the study research goals. These were divided into 16 publications of patients treated with SERM, of whom four had AI and four androgens. In general, the data regarding pharmacologic therapy for PG is partial, with insufficient evidence-based research. Tamoxifen and SERM drugs have long been used as treatments for PG. Tamoxifen was the chosen drug of treatment in most of the reviewed studies and found to be effective, safe, and with minimal side effects. CONCLUSIONS: Pharmacological treatment as a new standard of care has an advantage in relieving behavioral and psychological distress. Although high quality publications are lacking, pharmacological intervention with tamoxifen is appropriate in select patients. Conduction large-scale high-quality studies are warranted with various drugs.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9663914/

Record 642

pmcid: PMC8802657

pmid: 35057067

doi: 10.3390/pharmaceutics14010171

title: Herbal Medicines-Are They Effective and Safe during Pregnancy?

authors

- Sarecka-Hujar B
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journal: Pharmaceutics

journalAbbr: Pharmaceutics

year: 2022

pubdate: 2022 Jan 12

volume: 14

issue: 1

pages:

abstract: Since the teratogenicity of Thalidomide has been proven, herbal products are more commonly used in pregnancy to not only relieve morning sickness but also to fight infections. These products are frequently considered as natural and therefore harmless. However, herbs contain a number of active substances that, when used during pregnancy, can affect the development of the fetus. Often, pregnant women do not consult the usage of herbal medicines with a physician. The access to these products is easy and treatment of certain ailments with the use of herbs is common in many countries. The aim of the present literature review was to discuss available data regarding the efficacy and safety of cranberry, chamomile, Echinacea purpurea , garlic,

ginger, Ginkgo biloba , and peppermint, which are used to counteract the most common ailments during pregnancy, i.e., infections and pregnancy-related ailments (e.g., nausea and vomiting, dizziness, and headache). Analysis of available data showed that ginger is one of the most extensively analyzed herbal remedies. The dose of ginger below 1000 mg per day may help to relief hyperemesis gravidarum , and such an amount of ginger did not increase frequency of adverse effects for either woman or developing fetus. Data regarding other herbs are most often heterogeneous and give conflicting results with no clear conclusions. However, all herbal products should be used with a special caution in pregnancy. Further high-quality human studies should be determined to confirm the safe doses of herbal products which could be used by pregnant or breast-feeding women.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8802657/

Record 643

pmcid: PMC8781774

pmid: 35072069

doi: 10.1093/asjof/ojab046

title: High-Resolution Ultrasound-Assisted Assessment of Preliminary Short-term Safety Outcomes of an Implant-Based Augmentation Mammaplasty Using a Bioengineered, Cell-Friendly, Smooth-Surface Device in Korean Females.

authors

- Lee S
- Jeong JP
- Sung JY
- Choi WS
- Moon DS
- Kim HC
- Kim JH

journal: Aesthetic surgery journal. Open forum

journalAbbr: Aesthet Surg J Open Forum

year: 2022

pubdate: 2022

volume: 4

issue:

pages: ojab046

abstract: BACKGROUND: The Motiva Ergonomix Round SilkSurface (Establishment Labs Holdings Inc., Alajuela, Costa Rica) is one of the representative brands of the fifth generation of a silicone gel-filled breast implant with a microtextured surface. OBJECTIVES: In this study, the authors describe preliminary short-term safety outcomes of an implant-based augmentation mammaplasty using the Motiva Ergonomix Round SilkSurface in Korean females. METHODS: The authors performed a retrospective analysis of medical records in a total of 69 females (n = 69) receiving an implant-based augmentation mammaplasty using the Motiva Ergonomix Round SilkSurface between September 26, 2017, and December 31, 2020. The authors analyzed incidences of postoperative complications. RESULTS: A total of 6 cases (8.7%) of postoperative complications occurred; these include 2 cases (2.9%) of early seroma, 1 case (1.4%) of capsular contracture, 2 cases (2.9%) of alterations in the shape, and 1 case (1.4%) of foreign body sensation. Time-to-events were estimated at 266.81 ± 273.17 days. CONCLUSIONS: The authors describe our preliminary short-term safety outcomes of an implant-based augmentation mammaplasty using the Motiva Ergonomix Round SilkSurface in Korean females. But this deserves further large-scale studies with long periods of follow-up.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8781774/

Record 644

pmcid: PMC9889536

pmid: 36743932

doi: 10.3389/fendo.2022.1095950

title: Hormonal therapy is effective and safe for cryptorchidism caused by idiopathic hypogonadotropic hypogonadism in adult males.

authors

- Sun T
- Xu W
- Xu H
- Chen Y
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- Yang W
- Liu J

journal: Frontiers in endocrinology

journalAbbr: Front Endocrinol (Lausanne)

year: 2022

pubdate: 2022

volume: 13

issue:

pages: 1095950

abstract: BACKGROUND: Hormonal therapy is a reasonable treatment for cryptorchidism caused by idiopathic hypogonadotropic hypogonadism (IHH). However, the clinical evidence on whether it is effective and safe for the treatment of cryptorchidism caused by IHH is lacking. AIM: To evaluate the effect of hormonal therapy in testicular descent, puberty development, and spermatogenesis in adult males with cryptorchidism caused by IHH. METHODS: This retrospective study included 51 patients with cryptorchidism caused by IHH from the Andrology Clinic of University affiliated teaching hospital. Patients were divided into two groups: group A patients received hormonal therapy; group B patients received surgical treatment for cryptorchidism followed by hormonal therapy. RESULTS: The rate of successful testicular descent following hormonal therapy (19/32 in group A) or surgical treatment (11/19 in group B) shows no statistically significant difference. There was also no statistically significant difference in penile length, Tanner stage of pubic hair, testicular volume, and success rate of spermatogenesis between the two groups. Testicular atrophy was seen in a single patient in group B. CONCLUSIONS: Hormone therapy in adult males with cryptorchidism caused by IHH is effective and safe regarding testicular descent, puberty development, and spermatogenesis. This study provides new insight into the treatment of cryptorchidism caused by IHH and highlights that hormonal therapy could be an effective, safe, and economic treatment option for cryptorchidism in males caused by IHH.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9889536/

Record 645

pmcid: PMC8858793

pmid: 35223328

doi: 10.7759/cureus.22291

title: Hydatidiform Mole Presents As Pregnancy in a 48-Year-Old Perimenopausal Female: A Case Study.

authors

- Ftiha F
- Levada M
- Musheyev Y
- Garrick I
- Jiang M
- Ahasan H

journal: Cureus

journalAbbr: Cureus

year: 2022

pubdate: 2022 Feb

volume: 14

issue: 2

pages: e22291

abstract: Pregnancy should be suspected whenever a woman in her childbearing years misses a menstrual period. Clinical suspicion is increased if she also reports any sexual activity while not using contraception or is inconsistent in her use of contraception. Laboratory findings that aid in the diagnosis of pregnancy include the detection of human chorionic gonadotropin (hCG) in blood or urine. Hydatidiform mole (HM) is part of a group of diseases classified under gestational trophoblastic disease (GTD), which originate in the placenta and have the potential to locally invade the uterus and metastasize. Although molar pregnancies are designated as benign, they have the potential to develop into a malignancy. In this case study, we present a 48-year-old peri-menopausal female patient, with a 1+ year history of irregular menses, who presented to the clinic with signs and symptoms of pregnancy, unprotected sexual activity, and a positive at-home pregnancy test. Upon further workup of the patient, it was diagnosed that the patient had a hydatidiform molar pregnancy. It is interesting to note that benign gestational trophoblastic diseases generally occur in younger women, of "reproductive age" (generally in their twenties to early thirties), and is extremely rare in peri- and post-menopausal women.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8858793/

Record 646

pmcid: PMC9422229

pmid: 36017812

doi: 10.1530/EDM-22-0244

title: Idiopathic central precocious puberty with Prader-Willi syndrome: pubertal development with discontinuation of gonadotropin-releasing hormone analog.

authors

- Kobayashi M
- Yagasaki H
- Tamaru K
- Mitsui Y
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journal: Endocrinology, diabetes & metabolism case reports

journalAbbr: Endocrinol Diabetes Metab Case Rep

year: 2022

pubdate: 2022 Jan 1

volume: 2022

issue:

pages:

abstract: SUMMARY: Prader-Willi syndrome (PWS) is a genetic imprinting disorder that is characterized by obesity, short stature, and hypogonadism. Hypogonadism is characterized by normal luteinizing hormone (LH), high follicle-stimulating hormone (FSH), low testosterone, low inhibin B, and relatively low anti-Müllerian hormone (AMH). Only a few cases of central precocious puberty (CPP) have been reported in PWS, and follow-up for CPP with PWS is not established. Hence, we present a boy with PWS accompanied by CPP. Gonadotropin-releasing hormone analog (GnRHa) therapy was started at 7 years of age, CPP was adequately arrested, and GnRHa therapy was discontinued at 11.3 years of age. Growth hormone (GH) therapy was started at 12 years of age due to inadequate growth. He grew close to his final height, and his testes developed with normal LH, increased FSH, normal testosterone, and reduced AMH corresponding to puberty at 13.5 years of age. The features of 16 patients with PWS with CPP, including our patient, were summarized. Out of seven male patients, five were treated with GnRHa, as well as four out of nine female patients. Out of 16 patients, 6 were assessed with pubertal development over 13 years of age. Pubertal development was considered to be restored in four patients who had GnRHa therapy discontinuation. We should carefully follow-up on pubertal development in CPP. GnRHa therapy is useful for adequate puberty blockage, and pubertal development could be restored with GnRHa therapy discontinuation. LEARNING POINTS: Pubertal development in Prader-Willi syndrome (PWS) varies from hypogonadism to precocious puberty. Pubertal development assessment based on clinical features and hormone levels is needed in central precocious puberty (CPP) treatment with PWS. Gonadotropin-releasing hormone analog (GnRHa) therapy is useful for CPP with PWS, and pubertal development can be restored with GnRHa therapy discontinuation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9422229/

Record 647

pmcid: PMC8955082

pmid: 35350146

doi: 10.1097/GOX.0000000000004188

title: Immediate Breast Reconstruction in an 11-year-old Girl with a Large Malignant Phyllodes Tumor.

authors

- Serhan KA
- Kemp TL

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2022

pubdate: 2022 Mar

volume: 10

issue: 3

pages: e4188

abstract: Phyllodes tumors are rare fibroepithelial breast tumors representing less than 1% of all breast malignancies, with an extremely uncommon presence in the pediatric population. 1 Although prognosis is favorable following excision given their indolent course, they often grow rapidly and frequently recur. As such, they can present unique oncologic and reconstructive challenges. Herein we present a case of a malignant phyllodes tumor in an 11-year-old girl treated with total skin-sparing mastectomy and adjustable saline implant, and explore the reconstructive challenges of this unique case.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8955082/

Record 648

pmcid: PMC9762173

pmid: 36544865

doi: 10.2147/TCRM.S381800

title: Incidence and Predictors of Adverse Drug Events Among People Receiving Drug Resistant Tuberculosis Treatment in Uganda: 8-Year Retrospective Cohort Study.

authors

- Nasasira M
- Kalyango JN
- Mupere E
- Baluku JB

journal: Therapeutics and clinical risk management

journalAbbr: Ther Clin Risk Manag

year: 2022

pubdate: 2022

volume: 18

issue:

pages: 1117-1127

abstract: BACKGROUND: Adverse drug events (ADEs) are regarded as the most essential therapeutic issue during management of drug-resistant tuberculosis (DR-TB) due to the long duration of therapy and concurrent use of many second-line medications. This study aimed to determine the incidence and factors associated with ADEs among patients receiving DR-TB treatment at Mulago hospital in Uganda. METHODS: A retrospective cohort study was conducted among 417 DR-TB patient records at Mulago National Referral Hospital from January 2013 to December 2020. Using the data abstraction form, data were collected on socio-demographic and clinical factors, adverse drug events and treatment follow-up time. Data were double entered in Epi data version 3.2 and later exported to Stata version 14.0 for analysis. The incidence rate of adverse drug events was computed using number of cases of ADE divided by overall patient follow-up time. Poisson regression model was used to determine the factors associated with ADEs. The predictors were considered significant at if p< 0.05. RESULTS: The overall incidence was 5.56 ADEs per 100 person months (95% confidence interval (CI) 5.01, 6.15). Treatment regimens containing an aminoglycoside (incident rate ratio (IRR) 1.106, 95% CI 1.005-1.216 p=0.0391), linezolid (IRR 1.145, 95% CI 1.008-1.229 p = 0.037) or pyrazinamide (IRR 1.226, 95% CI 1.072-1.401 p = 0.003) and the treatment duration (in months) (IRR 1.005, 95% CI 1.001-1.010 p = 0.042) were associated with ADEs. CONCLUSION: Regimens containing aminoglycosides, linezolid, or pyrazinamide and increase in treatment duration (months) were associated with an increased risk of ADEs. Clinicians should quickly adopt all oral shorter treatment regimens to obviate the need for aminoglycosides and reduce exposure duration.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9762173/

Record 649

pmcid: PMC9117163

pmid: 35751199

doi: 10.1016/j.compbiomed.2022.105601

title: Insights into potential mechanisms of asthma patients with COVID-19: A study based on the gene expression profiling of bronchoalveolar lavage fluid.

authors

- Jiang Y
- Yan Q
- Liu CX
- Peng CW
- Zheng WJ
- Zhuang HF
- Huang HT
- Liu Q
- Liao HL
- Zhan SF
- Liu XH
- Huang XF

journal: Computers in biology and medicine

journalAbbr: Comput Biol Med

year: 2022

pubdate: 2022 Jul

volume: 146

issue:

pages: 105601

abstract: BACKGROUND: The 2019 novel coronavirus disease (COVID-19), caused by severe acute respiratory syndrome coronavirus 2 (SARS-CoV-2), is currently a major challenge threatening the global healthcare system. Respiratory virus infection is the most common cause of asthma attacks, and thus COVID-19 may contribute to an increase in asthma exacerbations. However, the mechanisms of COVID-19/asthma comorbidity remain unclear. METHODS: The "Limma" package or "DESeq2" package was used to screen differentially expressed genes (DEGs). Alveolar lavage fluid datasets of COVID-19 and asthma were obtained from the GEO and GSV database. A series of analyses of common host factors for COVID-19 and asthma were conducted, including PPI network construction, module analysis, enrichment analysis, inference of the upstream pathway activity of host factors, tissue-specific analysis and drug candidate prediction. Finally, the key host factors were verified in the GSE152418 and GSE164805 datasets. RESULTS: 192 overlapping host factors were obtained by analyzing the intersection of asthma and COVID-19. FN1, UBA52, EEF1A1, ITGB1, XPO1, NPM1, EGR1, EIF4E, SRSF1, CCR5, PXN, IRF8 and DDX5 as host factors were tightly connected in the PPI network. Module analysis identified five modules with different biological functions and pathways. According to the degree values ranking in the PPI network, EEF1A1, EGR1, UBA52, DDX5 and IRF8 were considered as the key cohost factors for COVID-19 and asthma. The H 2 O 2 , VEGF, IL-1 and Wnt signaling pathways had the strongest activities in the upstream pathways. Tissue-specific enrichment analysis revealed the different expression levels of the five critical host factors. LY294002, wortmannin, PD98059 and heparin might have great potential to evolve into therapeutic drugs for COVID-19 and asthma comorbidity. Finally, the validation dataset confirmed that the expression of five key host factors were statistically significant among COVID-19 groups with different severity and healthy control subjects. CONCLUSIONS: This study constructed a network of common host factors between asthma and COVID-19 and predicted several drugs with therapeutic potential. Therefore, this study is likely to provide a reference for the management and treatment for COVID-19/asthma comorbidity.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9117163/

Record 650

pmcid: PMC9023308

pmid: 35510137

doi: 10.3332/ecancer.2022.1356

title: International Society of Paediatric Surgical Oncology (IPSO) Surgical Practice Guidelines.

authors

- de Campos Vieira Abib S
- Chui CH
- Cox S
- Abdelhafeez AH
- Fernandez-Pineda I
- Elgendy A
- Karpelowsky J
- Lobos P
- Wijnen M
- Fuchs J
- Hayes A
- Gerstle JT

journal: Ecancermedicalscience

journalAbbr: Ecancermedicalscience

year: 2022

pubdate: 2022

volume: 16

issue:

pages: 1356

abstract: Most children with tumors will require one or more surgical interventions as part of the care and treatment, including making a diagnosis, obtaining adequate venous access, performing a surgical resection for solid tumors (with staging and reconstruction), performing procedures for cancer prevention and its late effects, and managing complications of treatment; all with the goal of improving survival and quality of life. It is important for surgeons to adhere to sound pediatric surgical oncology principles, as they are closely associated with improved local control and survival. Unfortunately, there is a significant disparity in survival rates in low and middle income countries, when compared to those from high income countries. The International Society of Paediatric Surgical Oncology (IPSO) is the leading organization that deals with pediatric surgical oncology worldwide. This organization allows experts in the field from around the globe to gather and address the surgical needs of children with cancer. IPSO has been invited to contribute surgical guidance as part of the World Health Organization Initiative for Childhood Cancer. One of our goals is to provide surgical guidance for different scenarios, including those experienced in High- (HICs) and Low- and Middle-Income Countries (LMICs). With this in mind, the following guidelines have been developed by authors from both HICs and LMICs. These have been further validated by experts with the aim of providing evidence-based information for surgeons who care for children with cancer. We hope that this initiative will benefit children worldwide in the best way possible. Simone Abib, IPSO President Justin T Gerstle, IPSO Education Committee Chair Chan Hon Chui, IPSO Secretary.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9023308/

Record 651

pmcid: PMC9556436

pmid: 36224187

doi: 10.1038/s41467-022-33826-4

title: Key considerations to reduce or address respondent burden in patient-reported outcome (PRO) data collection.

authors

- Aiyegbusi OL
- Roydhouse J
- Rivera SC
- Kamudoni P
- Schache P
- Wilson R
- Stephens R
- Calvert M

journal: Nature communications

journalAbbr: Nat Commun

year: 2022

pubdate: 2022 Oct 12

volume: 13

issue: 1

pages: 6026

abstract: Patient-reported outcomes (PROs) are used in clinical trials to provide evidence of the benefits and risks of interventions from a patient perspective and to inform regulatory decisions and health policy. The collection of PROs in routine practice can facilitate monitoring of patient symptoms; identification of unmet needs; prioritisation and/or tailoring of treatment to the needs of individual patients and inform value-based healthcare initiatives. However, respondent burden needs to be carefully considered and addressed to avoid high rates of missing data and poor reporting of PRO results, which may lead to poor quality data for regulatory decision making and/or clinical care.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9556436/

Record 652

pmcid: PMC8901272

pmid: 35265320

doi: 10.1093/jscr/rjac067

title: Late infection of a breast prosthesis with staphylococcus aureus in a healthy woman: a case report.

authors

- Dartaha R
- Brimo Alsaman MZ
- Jobran AWM

journal: Journal of surgical case reports

journalAbbr: J Surg Case Rep

year: 2022

pubdate: 2022 Mar

volume: 2022

issue: 3

pages: rjac067

abstract: Infection following breast augmentation is a rare event with an incidence rate of 1-2.5%. Late onset

infections following breast augmentation are very rare. Herein, we present a case of breast implant infection in a 29-year-old female patient who underwent a bilateral augmentation mammoplasty with a silicone gel prosthesis. After 8 uneventful post-operative years, she presented with right-sided signs of breast infection. She initially treated medically but without improvement. Then, she underwent surgical washout and debridement with removal of the bilateral breast implants. Culture demonstrated Staphylococcus aureus . The clinical history and management of this unusual case are described. Because most of the infections occurs in the first few weeks after augmentation mammoplasty, there is a paucity of data about late onset infections. The most common cultured organism in the early infection is S. aureus .

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8901272/

Record 653

pmcid: PMC9615954

pmid: 36018205

doi: 10.4103/ajps.ajps_92_21

title: Mastitis and breast abscess in newborns and infants.

authors

- Jean Bertrand KA
- Rose NK
- Franck LG
- Célestin BA
- Ibrahim T
- N'gouan Constance BU

journal: African journal of paediatric surgery : AJPS

journalAbbr: Afr J Paediatr Surg

year: 2022

pubdate: 2022 Oct-Dec

volume: 19

issue: 4

pages: 238-240

abstract: INTRODUCTION: Mastitis and abscess of the newborn breast are serious infections. The objective of our study was to describe the epidemiological and therapeutic aspects of these infections in our context. MATERIALS AND METHODS: This was a retrospective study from January 2017 to December 2019. We included all children aged ≤2 months admitted for mastitis or breast abscess. The variables studied were epidemiological and therapeutic. RESULTS: We collected 34 files, i.e., 11.3 cases/year. The sex ratio was 0.5. The mean age was 16.3 ± 6.6 days. The mean time to the consultation was 3.7 ± 2.4 days. The parents had undertaken a massage in 79.4% of cases. It was mastitis in 12 cases and abscess in 21 cases. Treatment was surgical in 26 cases. CONCLUSION: This affection is frequent with a female predominance. Its management must be early. The proscription of breast massage would reduce its frequency.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9615954/

Record 654

pmcid: PMC9205636

pmid: 35721320

doi: 10.3389/fnmol.2022.892477

title: Metformin in the Treatment of Amisulpride-Induced Hyperprolactinemia: A Clinical Trial.

authors

- Zhu C
- Li R
- Ju M
- Xiao X
- Yuan TF
- Jin Z
- Zhao J

journal: Frontiers in molecular neuroscience

journalAbbr: Front Mol Neurosci

year: 2022

pubdate: 2022

volume: 15

issue:

pages: 892477

abstract: OBJECTIVE: To evaluate the efficacy and safety of metformin in the treatment of amisulpride-induced hyperprolactinemia. METHODS: A total of 86 schizophrenic patients who developed hyperprolactinemia after taking amisulpride were screened and randomly assigned to the metformin group (42 patients) and placebo group (44 patients) and followed up for eight weeks. The patients' serum prolactin levels, blood glucose and lipids were measured at the baseline and the end of the intervention. The treatment emergent symptom scale (TESS) was also assessed. RESULTS: After eight weeks of intervention, serum prolactin levels in the metformin group decreased from (1737.360 ± 626.918) mIU/L at baseline to (1618.625 ± 640.865) mIU/L, whereas serum prolactin levels in the placebo group increased from (2676.470 ± 1269.234) mIU/L at baseline to (2860.933 ± 1317.376) mIU/L. There was a significant difference in prolactin changes (F covariance = 9.982, P = 0.002) between the two groups. There was no significant difference in the incidence of adverse drug reactions (P > 0.05) between the two groups. CONCLUSION: Metformin is able to improve amisulpride-induced hyperprolactinemia with its safety.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9205636/

Record 655

pmcid: PMC9112044

pmid: 35591886

doi: 10.2147/NDT.S348143

title: Minimal Effects of Cariprazine on Prolactin Levels in Bipolar Disorder and Schizophrenia.

authors

- Culpepper L
- Vieta E
- Kelly DL
- Patel MD
- Szatmári B
- Hankinson A
- Earley WR

journal: Neuropsychiatric disease and treatment

journalAbbr: Neuropsychiatr Dis Treat

year: 2022

pubdate: 2022

volume: 18

issue:

pages: 995-1011

abstract: BACKGROUND: Many medications used to treat schizophrenia and bipolar I disorder are linked to hyperprolactinemia. The effects of cariprazine, a dopamine D 3 /D 2 receptor partial agonist, on prolactin levels in patients with schizophrenia or bipolar I disorder were evaluated. METHODS: Effects on prolactin were evaluated using pooled data from randomized, double-blind, placebo-controlled studies in patients with schizophrenia (4 studies; 6-week duration; cariprazine 1.5-3 mg/d, 4.5-6 mg/d, and 9-12 mg/d), bipolar mania (3 studies; 3-week duration; cariprazine 3-6 and 9-12 mg/d), and bipolar depression (3 studies; 6- to 8-week duration; cariprazine 1.5 and 3 mg/d). Long-term effects were analyzed using open-label studies in patients with schizophrenia (2 studies; 48-week duration) and patients with bipolar mania (1 study; 16-week duration). Change in prolactin levels (ng/mL) from baseline to study endpoint was evaluated in subsets of sex and prior medication use. RESULTS: In patients with schizophrenia (male, n = 1377; female, n = 558), median prolactin changes were -1.2 for males and -7.4 for females on placebo, and ranged from -4.2 to -3.6 for males and -12.4 to +0.2 for females in the cariprazine-treatment groups. In patients with bipolar mania (male, n = 570; female, n = 395), median prolactin changes were -0.2 for males and -1.1 for females on placebo and ranged from -2.1 to -3.0 for males and 0 to +1.8 for females in the cariprazine-treatment groups. Median decreases were also seen in the long-term studies of schizophrenia (range, -14.6 to -2.0) and bipolar mania (range, -0.8 to +1.9). In patients with bipolar depression (male, n = 485; female, n = 780), median prolactin changes were +0.3 for males and +0.7 for females on placebo and ranged from +0.4 to +0.5 for males and +3.0 to +3.1 for females in the cariprazine-treatment groups. CONCLUSION: Treatment with cariprazine for schizophrenia or bipolar I disorder was associated with minimal effects on prolactin levels.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9112044/

Record 656

pmcid: PMC9658829

pmid: 36364082

doi: 10.3390/molecules27217257

title: Modification of Cotton Fabric with Molecularly Imprinted Polymer-Coated Carbon Dots as a Sensor for 17 α-methyltestosterone.

authors

- Lim M
- Thanasupsin SP
- Thongkon N

journal: Molecules (Basel, Switzerland)

journalAbbr: Molecules

year: 2022

pubdate: 2022 Oct 26

volume: 27

issue: 21

pages:

abstract: Molecularly imprinted polymers@ethylenediamine-modified carbon dots grafted on cotton fabrics (MIPs@EDA-CDs/CF) and smartphone-based fluorescence image analysis were proposed and used for the first time for the detection of 17 α-methyltestosterone (MT). The EDA-CDs were synthesized and grafted on cotton

fabric before coating with the MIPs. The MIPs were synthesized using the MT as a template molecule, methacrylic acid (MAA) as a functional monomer, ethylene glycol dimethacrylate (EGDMA) as a cross-linker, and azobisisobutyronitrile (AIBN) as an initiator. The MIPs@EDA-CDs/CF were characterized using FTIR, SEM-EDS, and RGB fluorescence imaging. The fluorescence images were also taken using a smartphone and the ImageJ program was used for RGB measurement. The Δ red intensity was linearly proportional to MT concentration in the range of 100 to 1000 μg/L (R 2 = 0.999) with a detection limit of 44.4 μg/L and quantification limit of 134 μg/L. The MIPs@EDA-CDs/CF could be stored at 4 °C for a few weeks and could be reused twice. The proposed method could apply for the specific determination of MT in water and sediment samples along with satisfactory recoveries of 96-104% and an acceptable relative standard deviation of 1-6% at the ppb level.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9658829/

Record 657

pmcid: PMC8884531

pmid: 35242490

doi: 10.1097/GOX.0000000000004138

title: Muscle Splitting Augmentation Mastopexy: A l3-year Analysis and Outcome of Primary and Secondary Procedures.

authors

- Khan UD

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2022

pubdate: 2022 Feb

volume: 10

issue: 2

pages: e4138

abstract: BACKGROUND: The muscle splitting technique is a submuscular pocket used for augmentation mammoplasty. Its use and results following primary and secondary procedures are presented. METHODS: A retrospective study that analyzed complication and revision rates following primary and secondary mastopexy with augmentation in a muscle splitting pocket performed between April 2007 and March 2020 was carried out. Data were divided into two groups: primary and secondary single-stage mastopexies with augmentation. RESULTS: A total of 169 patients had a primary procedure and 49 had secondary procedures. The mean ages of patients in the primary and secondary were 32.3 ± 9.89 and 38.2 ± 9.63 years, respectively. Five (3.0%) patients had periprosthetic or wound infections in the primary group when compared with 0% in secondary surgery group. Wound breakdown was seen in 13 (7.7%) patients with a primary procedure compared with one patient (2%) with a secondary procedure. One patient had partial nipple loss and another had total nipple loss in the primary group (1.2%) when compared with one patient (2.0%) who had partial nipple loss in the secondary group. Revision surgeries were carried out in 16 (10.1%) of the primary procedures compared with five (10.2%) of the secondary procedures. CONCLUSION: Even though there was a higher prevalence of complications in the primary procedures group than in the secondary procedures group, the overall complications following muscle splitting mastopexy with augmentation are within an acceptable range.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8884531/

Record 658

pmcid: PMC9592347

pmid: 36281154

doi: 10.1097/MD.0000000000031146

title: Network-based pharmacological study of the mechanism of kaiyuzhongyutang in the treatment of tubal fimbria obstruction.

authors

- Li Z
- Liu X
- Wang S
- Han L
- Chen P
- Zhong T
- Wang B

journal: Medicine

journalAbbr: Medicine (Baltimore)

year: 2022

pubdate: 2022 Oct 21

volume: 101

issue: 42

pages: e31146

abstract: To apply a network pharmacological approach to explore the targets and possible mechanisms of Kai Yu Zhong Yu Tang (KYZYT) in the treatment of tubal fimbria obstruction. The target information of KYZYT was extracted from TCMSP and HERB database. Genes related to tubal fimbria obstruction were searched using the GENECARD database. Target protein network maps (PPI) were drawn using string database analysis and Cytoscape 3.7.1 software. Kyoto Encyclopedia of Genes and Genomes (KEGG) pathway enrichment analysis and gene function analysis (GO) enrichment analysis were performed with the help of Perl language and biological program package in R language. To explore the multiple pharmacological mechanisms of action of KYZYT in the interventional treatment of tubal fimbria obstruction and to lay the foundation for further experimental validation. Through the collection and analysis of multiple databases, 355 biological targets of KYZYT were identified. 168 targets of tubal fimbria obstruction were obtained from disease database. The "drug-component" and "drug-target" networks of KYZYT were constructed, and the protein interaction network (PPI) of overlapping targets was analyzed to identify the key targets of the drug affecting the disease. In addition, KEGG pathway analysis and GO enrichment analysis were performed on the overlapping targets to explore the mechanism of KYZYT in the treatment of tubal fimbria obstruction. KYZYT has the characteristics of multi-component, multi-target and multi-pathway in the treatment of tubal fimbria obstruction, which provides new ideas and scientific basis for further clarification of the molecular mechanism.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9592347/

Record 659

pmcid: PMC8734530

pmid: 35059599

doi: 10.4274/ejbh.galenos.2020.5541

title: Nipple Eczema Causing Galactorrhea by Reactive Hyperprolactinemia, Complicated by a Galactocele.

authors

- Chassain K
- Vrignaud A
- Cesbron E
- Bara-Passot C

- Maillard H

journal: European journal of breast health

journalAbbr: Eur J Breast Health

year: 2022

pubdate: 2022 Jan

volume: 18

issue: 1

pages: 102-104

abstract: We present a case of atopic nipple eczema leading to reactive hyperprolactinemia, by mechanical nipple stimulation. This reactive hyperprolactinemia caused an aggravation of the eczema because of the resulting galactorrhea, by local irritation and inflammation, and was complicated by a galactocele. This benign tumour was a source of concern for the patient and required several diagnostic radiographic examinations.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8734530/

Record 660

pmcid: PMC9624624

pmid:

doi: 10.1210/jendso/bvac150.356

title: ODP113 Presentation of two similar Klinefelter Syndrome cases with different clinical outcomes.

authors

- Marenych N
- Moid A
- Trendafilova V
- Gilden JL

journal: Journal of the Endocrine Society

journalAbbr: J Endocr Soc

year: 2022

pubdate: 2022 Nov 1

volume: 6

issue: Suppl 1

pages: A173-4

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9624624/

Record 661

pmcid: PMC9383022

pmid: 35848651

doi: 10.4103/aam.aam_93_20

title: Ovarian cell tumor in a child with neurofibromatosis type 1.

authors

- Qutub LM

- Al-Agha AE

  journal: Annals of African medicine

  journalAbbr: Ann Afr Med

  year: 2022

  pubdate: 2022 Apr-Jun

  volume: 21

  issue: 2

  pages: 165-167

  abstract: Juvenile granulosa cell ovarian tumor is a rare cause ofpseudo-precociouspuberty. We report a case of a 6-year-old female with neurofibromatosis type 1 (NF1), associated with pseudo-precocious puberty (PPP). A thorough workup revealed a large multi-cystic right ovarian mass, which turned out to be a juvenile granulosa cell tumor (JGCT). This report documented a rare case of PPP caused by JGCT in a child with NF1. Verbal consent was taken from the family.

  url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9383022/

Record 662

pmcid: PMC10358864

pmid: 37483405

doi: 10.21980/J8035H

title: Ovarian Juvenile Granulosa Cell Tumor Case Report.

authors

- Lemmons J
- Little-Wienert K
- Hamad A

  journal: Journal of education & teaching in emergency medicine

  journalAbbr: J Educ Teach Emerg Med

  year: 2022

  pubdate: 2022 Jan

  volume: 7

  issue: 1

  pages: V8-V12

  abstract: UNLABELLED: A 5-month-old female presented to the emergency department with worsening abdominal distention and postprandial emesis. Point-of-care ultrasound (POCUS) demonstrated extensive abdominal free fluid with a large, heterogeneous mass in the suprapubic region. This was confirmed on computed tomography (CT) of the abdomen and pelvis. The patient was urgently taken to the operating room for exploratory laparotomy where a ruptured tumor was discovered. The patient was eventually diagnosed with ovarian juvenile granulosa cell tumor. POCUS can be vital in guiding the recognition of rare pediatric conditions in the emergency department and should be considered in patients where there is concern for an acute abdomen. TOPICS: Abdominal pain, ascites, ovarian juvenile granulosa cell tumor, point-of-care ultrasound.

  url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10358864/

Record 663

pmcid: PMC9082855

pmid: 35527271

doi: 10.1186/s12978-022-01408-8

title: Parental communication on sexual and reproductive health issues to their adolescents and affecting factors at Asella town, Ethiopia: a community-based, cross-sectional study.

authors

- Bekele D
- Deksisa A
- Abera W
- Megersa G

journal: Reproductive health

journalAbbr: Reprod Health

year: 2022

pubdate: 2022 May 8

volume: 19

issue: 1

pages: 114

abstract: BACKGROUND: Parents' communication on sexual and reproductive health issues with their adolescent plays a great role in preventing morbidity and mortality associated with sexual behavior. However lack of parent to adolescent communication was a serious problem in Ethiopia resulted in teenage pregnancy, unsafe abortions, sexually transmitted infections, school problems, and other sexual risk behaviors. Parents have high responsibility on cultivating their son and daughter regarding to sexual and reproductive health issues. Therefore, the objective of this study was to determine the magnitude of parent's communication with their adolescents and affecting factors in Ethiopia. METHODS: A community-based, cross-sectional study was conducted on 347 respondents. A systematic sampling method was used to select the study participants. Data were collected by trained interviewers using a structured questionnaire, entered into Epi-Info version 7.1.2 and exported to SPSS version 23 for analysis. Descriptive statistics, bivariate and multivariate logistic regression analyses were used. Variables at P-value < 0.05 were considered as significant associations. RESULTS: Slightly more than one-fifth of the parents (21.3%) had communicated with their adolescents on sexual and reproductive health issues. Associated factors like: being knowledgeable [AOR = 3.08, 95% CI: 1.89-5.39] and being having positive attitudes [AOR 3.03, 95% CI: 1.37-6.70] towards sexual reproductive health issues were significantly associated with communication. CONCLUSION: Overall a low proportion of parental communication with their children was identified on sexual and reproductive health issues. This was affected by multidimensional factors to determine their discussion. Thus, promotion of parent to adolescent communication, parents training and addressing the importance of parent to young people communication along with health care providers was important.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9082855/

Record 664

pmcid: PMC8882713

pmid: 34463715

doi: 10.1093/cid/ciab719

title: Pathology and Pathogenesis of Lassa Fever: Novel Immunohistochemical Findings in Fatal Cases and Clinico-pathologic Correlation.

authors

- Shieh WJ
- Demby A
- Jones T

- Goldsmith CS
- Rollin PE
- Ksiazek TG
- Peters CJ
- Zaki SR

journal: Clinical infectious diseases : an official publication of the Infectious Diseases Society of America

journalAbbr: Clin Infect Dis

year: 2022

pubdate: 2022 May 30

volume: 74

issue: 10

pages: 1821-1830

abstract: BACKGROUND: Lassa fever is a zoonotic, acute viral illness first identified in Nigeria in 1969. An estimate shows that the "at risk" seronegative population (in Sierra Leone, Guinea, and Nigeria) may be as high as 59 million, with an annual incidence of all illnesses of 3 million, and fatalities up to 67 000, demonstrating the serious impact of the disease on the region and global health. METHODS: Histopathologic evaluation, immunohistochemical assay, and electron microscopic examination were performed on postmortem tissue samples from 12 confirmed Lassa fever cases. RESULTS: Lassa fever virus antigens and viral particles were observed in multiple organ systems and cells, including cells in the mononuclear phagocytic system and other specialized cells where it had not been described previously. CONCLUSIONS: The immunolocalization of Lassa fever virus antigens in fatal cases provides novel insightful information with clinical and pathogenetic implications. The extensive involvement of the mononuclear phagocytic system, including tissue macrophages and endothelial cells, suggests participation of inflammatory mediators from this lineage with the resulting vascular dilatation and increasing permeability. Other findings indicate the pathogenesis of Lassa fever is multifactorial and additional studies are needed.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8882713/

Record 665

pmcid: PMC9142250

pmid: 35832157

doi: 10.1055/s-0042-1748649

title: Patient-Reported Outcomes after Subpectoral Breast Augmentation with Microtextured or Macrotextured Implants Using the BREAST-Q.

authors

- Leite AT
- Sabino-Neto M
- Resende VCL
- Veiga DF
- Ferreira LM

journal: Archives of plastic surgery

journalAbbr: Arch Plast Surg

year: 2022

pubdate: 2022 May

volume: 49

issue: 3

pages: 352-359

abstract: Background Breast augmentation with implants is the most commonly performed cosmetic plastic surgery in Brazil and worldwide. The aim of this study was to assess patient satisfaction and quality of life following subpectoral breast augmentation with either microtextured or macrotextured implants, using the BREAST-Q. Methods A prospective study was conducted with 40 women with hypomastia undergoing subpectoral breast augmentation. The patients were randomly allocated to two groups to receive either microtextured or macrotextured breast implants. All participants were assessed preoperatively (baseline) and after 2 and 4 months of surgery for quality of life and patient satisfaction with the surgical results, using the BREAST-Q augmentation module, a patient-reported outcome measure. Results The patients had a mean age of 28.9 ± 6.45 years. The microtextured (n = 20) and macrotextured (n = 20) groups were homogeneous for sex, age, education level, marital status, and number of children (p > 0.05). Both groups showed significant improvement in satisfaction with breasts (p < 0.001), psychosocial well-being (p < 0.001), and sexual well-being (p < 0.001) at the 2- and 4-month follow-up visits compared with baseline. The observed improvements were associated with high effect size values of 5.09, 3.44, and 3.90, respectively. In contrast, significant decreases from baseline in physical well-being scores (p = 0.001) were found 2 and 4 weeks after surgery in both groups. Conclusion Subpectoral breast augmentation with either microtextured or macrotextured breast implants improved satisfaction with breasts and quality of life in patients with hypomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9142250/

Record 666

pmcid: PMC8803227

pmid: 35136635

doi: 10.1259/bjrcr.20210077

title: Pediatric breast lymphatic malformation with recurrent presentation in an adolescent female.

authors

- Kaye R
- Leddy R

journal: BJR case reports

journalAbbr: BJR Case Rep

year: 2022

pubdate: 2022 Jan 1

volume: 8

issue: 1

pages: 20210077

abstract: This case report summarizes a rare case of left chest wall/breast lymphatic malformation or cystic lymphangioma in a female child of 18 months with multiple late recurrences in adolescence. By maternal history, the mass was excised initially, but the patient presented at age 15 and 17 years for recurrences and associated symptoms. Comments focus on a complex clinical history and treatment management of patient symptoms and concerns. Breast sparing treatments were employed with sclerotherapy and the T lymphocyte inhibitor, Sirolimus (Rapamune).

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8803227/

Record 667

pmcid: PMC9805112

pmid: 35883220

doi: 10.1111/joim.13547

title: Pediatric obesity-Long-term consequences and effect of weight loss.

authors

- Marcus C
- Danielsson P
- Hagman E

journal: Journal of internal medicine

journalAbbr: J Intern Med

year: 2022

pubdate: 2022 Dec

volume: 292

issue: 6

pages: 870-891

abstract: Childhood obesity is, according to the WHO, one of the most serious challenges of the 21st century. More than 100 million children have obesity today. Already during childhood, almost all organs are at risk of being affected by obesity. In this review, we present the current knowledge about diseases associated with childhood obesity and how they are affected by weight loss. One major causative factor is obesity-induced low-grade chronic inflammation, which can be observed already in preschool children. This inflammation-together with endocrine, paracrine, and metabolic effects of obesity-increases the long-term risk for several severe diseases. Type 2 diabetes is increasingly prevalent in adolescents and young adults who have had obesity during childhood. When it is diagnosed in young individuals, the morbidity and mortality rate is higher than when it occurs later in life, and more dangerous than type 1 diabetes. Childhood obesity also increases the risk for several autoimmune diseases such as multiple sclerosis, Crohn's disease, arthritis, and type 1 diabetes and it is well established that childhood obesity also increases the risk for cardiovascular disease. Consequently, childhood obesity increases the risk for premature mortality, and the mortality rate is three times higher already before 30 years of age compared with the normal population. The risks associated with childhood obesity are modified by weight loss. However, the risk reduction is affected by the age at which weight loss occurs. In general, early weight loss-that is, before puberty-is more beneficial, but there are marked disease-specific differences.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9805112/

Record 668

pmcid: PMC9742306

pmid: 35867279

doi: 10.1007/s40572-022-00366-4

title: Phenols, Parabens, Phthalates and Puberty: a Systematic Review of Synthetic Chemicals Commonly Found in Personal Care Products and Girls' Pubertal Development.

authors

- Rivera-Núñez Z
- Kinkade CW
- Zhang Y
- Rockson A
- Bandera EV
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- Barrett ES

journal: Current environmental health reports

journalAbbr: Curr Environ Health Rep

year: 2022

pubdate: 2022 Dec

volume: 9

issue: 4

pages: 517-534

abstract: PURPOSE OF REVIEW: Exposure to endocrine disrupting chemicals through personal care products (PCPs) is widespread and may disrupt hormone-sensitive endpoints, such as timing of puberty. Given the well-documented (and ongoing) decline in age at menarche in many populations, we conducted a systematic review of the epidemiological literature on exposure to chemicals commonly found in PCPs (including certain phthalates, phenols, and parabens) in relation to girls' pubertal development. RECENT FINDINGS: The preponderance of research on this topic has examined phthalate exposures with the strongest evidence indicating that prenatal monoethyl phthalate (MEP) concentrations may be associated with slightly earlier timing of puberty, including age at menarche. Findings examining peri-pubertal phthalate exposures and pubertal outcomes were less consistent as were studies of prenatal and peri-pubertal phenol exposures. Very few studies had examined parabens in relation to girls' pubertal development. Common study limitations included potential exposure misclassification related to use of spot samples and/or mistimed biomarker assessment with respect to the outcomes. The role of body size as a mediator in these relationships remains unresolved. Overall, evidence of associations between chemical exposures in PCPs and girls' pubertal development was conflicting. When associations were observed, effect sizes were small. Nevertheless, given the many environmental, social, and behavioral factors in the modern environment that may act synergistically to accelerate timing of puberty, even marginal changes may be cause for concern, with implications for cancer risk, mental health, and cardiometabolic disease in later life.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9742306/

Record 669

pmcid: PMC9281080

pmid: 35836205

doi: 10.1186/s12887-022-03474-0

title: Precocious puberty or growth hormone deficiency as initial presentation in Mayer-Rokitansky-kuster-Hauser syndrome: a clinical report of 5 cases.

authors

- Ai Z
- Zhu X
- Chen H
- Chen R

journal: BMC pediatrics

journalAbbr: BMC Pediatr

year: 2022

pubdate: 2022 Jul 14

volume: 22

issue: 1

pages: 418

abstract: BACKGROUND: We report five patients with Mayer-Rokitansky-Küster-Hauser syndrome (MRKHS),

four of whom presented with precocious puberty and one with growth hormone deficiency (GHD. Our five children add to the growing endocrine data base of MRKHS. CASE PRESENTATION: We retrospectively reviewed clinical data of 5 MRKHS patients from 2017 to 2020. The clinical features, hormonal profiles, radiological imaging and genetic analyses were collated. The age range of the 5 patients at diagnosis was 6.7-9.1 years. Four presented with premature thelarche, and one presented with short stature. External genitalia were normal in all patients. Gonadotropin-releasing hormone stimulation tests for the 5 patients revealed peak luteinizing hormone and follicular stimulating hormone levels of 3.57, 6.24, 11.5, 4.44 and 4.97 IU/L and 9.41, 16.7, 13.8, 14.2 and 10.3 mIU/mL, respectively. Growth hormone stimulation for one patient with short stature was consistent with GHD with a peak level of GH was 7.30 ng/mL. Imaging disclosed advanced bone age in four patients and no skeletal abnormalities in any of the patients. Ultrasonography of the abdomen revealed bilateral polycystic kidneys in one patient. Pelvic magnetic resonance imaging confirmed no uterus in five patients. All of the patients had a normal karyotype (46, XX). In one patient, whole-exome sequencing detected a deletion of 17q12(chr17:36,046,434-36,105,050, hg19) encompassing the HNF1B gene. CONCLUSIONS: We report the unusual co-occurrence of precocious puberty and GHD in patients with MRKHS, highlighting that abnormal puberty and growth development may represent initial unexplained manifestations. Whether the deletion of 17q 22 begat GHD is unclear.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9281080/

Record 670

pmcid: PMC9382462

pmid: 36032464

doi: 10.4314/ahs.v22i1.10

title: Premenstrual syndrome: consultation sources and the impact on women's quality of life.

authors

- Jaber RM
- Alghzawi AO
- Salameh HH

journal: African health sciences

journalAbbr: Afr Health Sci

year: 2022

pubdate: 2022 Mar

volume: 22

issue: 1

pages: 80-87

abstract: INTRODUCTION: The main aim of this study was to explore the sources of consultation that women seek during premenstural syndrome (PMS), and to establish the association between the severity of PMS and how it affects the quality of life. METHODS: Cross sectional study of 179 female patients, using an Arabic validated version of the shortened premenstrual assessment form, and a 7-item quality of life questionnaire that was developed by the researchers. The questionnaire was tested for validity and reliability via a pilot study before the initiation of data collection. Data was analyzed using SPSS version 19. RESULTS: 179 women participated in the study, with more than half of them aged between 20-30 years old. PMS prevalence was 88%; patient's predominantly sought help from their relatives (51%), followed by physicians (34%). There was no association found between the severity of premenstrual symptoms and seeking consultation. PMS symptoms affected women's daily activities (p-value 0.039), their satisfaction with their general appearance (p-value 0.001) and weight (p-value 0.022), and affected their relationships with family members (p-value 0.001) and other people (p-value 0.002). CONCLUSION: PMS is a common condition that affects women and their quality of life in several ways. Physicians and primary health care providers must be more vigilant in detecting the presence of PMS.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9382462/

Record 671

pmcid: PMC9434240

pmid: 35466359

doi: 10.1093/humupd/dmac013

title: Prenatal and postnatal exposures to endocrine disrupting chemicals and timing of pubertal onset in girls and boys: a systematic review and meta-analysis.

authors

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- Lim YH
- Gregersen LS
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journal: Human reproduction update

journalAbbr: Hum Reprod Update

year: 2022

pubdate: 2022 Aug 25

volume: 28

issue: 5

pages: 687-716

abstract: BACKGROUND: Globally, the ages at pubertal onset for girls and boys have been decreasing during recent decades, partly attributed to excess body fat accumulation. However, a growing body of literature has recognized that endocrine disrupting chemicals (EDCs) may play an important role in this global trend, but the association has not yet been fully established. OBJECTIVE AND RATIONALE: EDCs can interfere with normal hormone function and metabolism and play a role in pubertal onset. We aimed to systematically identify and evaluate the current evidence on the timing of pubertal onset in girls and boys following prenatal or postnatal exposures to xenobiotic EDCs. SEARCH METHODS: Following PRISMA guidelines, we performed a systematic literature search of original peer-reviewed publications in the PubMed database through a block search approach using a combination of index MeSH and free text search terms. Publications were considered if they covered biomarkers of prenatal or postnatal exposures to xenobiotic EDCs (European Commission's list of category 1 EDCs) measured in maternal or child biospecimen and pubertal onset defined by the progression of the following milestones (and assessed in terms of the following measures): menarche (age), thelarche (Tanner staging) and pubarche (Tanner staging), in girls, and genital stage (Tanner staging), testicular volume (ml) and pubarche (Tanner staging), in boys. OUTCOMES: The literature search resulted in 703 references, of which we identified 52 publications fulfilling the eligibility criteria for the qualitative trend synthesis and 23 publications for the meta-analysis. The qualitative trend synthesis provided data on 103 combinations of associations between prenatal or postnatal exposure to EDC compounds groups and puberty outcomes and the meta-analysis enabled 18 summary risk estimates of meta-associations. WIDER IMPLICATIONS: Statistically significant associations in the qualitative trend synthesis suggested that postnatal exposure to phthalates may be associated with earlier thelarche and later pubarche. However, we did not find consistent evidence in the meta-

analysis for associations between timing of pubertal onset in girls and boys and exposures to any of the studied xenobiotic EDCs. We were not able to identify specific pre- or postnatal windows of exposure as particularly critical and susceptible for effects of EDCs. Current evidence is subject to several methodological challenges and inconsistencies and evidence on specific exposure-outcome associations remains too scarce to firmly confirm EDC exposure as a risk factor for changes in age of pubertal onset in the general child population. To create a more uniform foundation for future comparison of evidence and to strengthen pooled studies, we recommend the use of more standardized approaches in the choice of statistical analyses, with exposure transformations, and in the definitions and assessments of puberty outcomes. The impact of mixtures of EDC exposures on the association also remains unestablished and would be valuable to elucidate for prenatal and postnatal windows of exposure. Future large, longitudinal epidemiological studies are needed to clarify the overall association.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9434240/

Record 672

pmcid: PMC8987598

pmid: 35402679

doi: 10.1016/j.jpra.2022.01.004

title: Primary Lymphomas of the Breast: A Review.

authors

- James ER
- Miranda RN
- Turner SD

journal: JPRAS open

journalAbbr: JPRAS Open

year: 2022

pubdate: 2022 Jun

volume: 32

issue:

pages: 127-143

abstract: Lymphomas of the breast are rare neoplasms that arise from breast lymphoid tissue and are characterised by neoplastic B or T cells. Breast lymphomas arising from B cells include, but are not limited to, diffuse large B cell lymphoma, follicular lymphoma, extra-nodal marginal zone lymphoma and Burkitt lymphoma. Anaplastic large cell lymphoma (ALCL) is of a T cell origin and both anaplastic lymphoma kinase (ALK)-positive and ALK-negative presentations have been noted in the breast. In addition, there is a more recently identified presentation of ALK-negative ALCL that arises around textured breast implants and is usually confined to a periprosthetic fibrous capsule. Here, we discuss the clinical presentations, histological and immunohistochemical features and treatment options for each type of primary breast lymphoma. We hope that this review will highlight the importance of the timely and accurate diagnosis of breast lymphoma in order to tailor the most appropriate treatment. We also wish to raise awareness of the breast implant-associated lymphomas, with the goal of stimulating work that will aid our understanding of their epidemiology and pathogenesis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8987598/

Record 673

pmcid: PMC10558402

pmid: 35721880

doi: 10.4331/wjbc.v13.i3.66

title: Progesterone in gender-affirming therapy of trans women.

authors

- Milionis C
- Ilias I
- Koukkou E

journal: World journal of biological chemistry

journalAbbr: World J Biol Chem

year: 2022

pubdate: 2022 May 27

volume: 13

issue: 3

pages: 66-71

abstract: Progesterone is an endogenous steroid hormone with an important role for the physiology of the female reproductive system and the mammary gland. It has additional significant actions in other tissues, such as the cardiovascular system, the central nervous system, and bones. The present article explores potential clinical implications from the addition of bioidentical progesterone to gender-affirming treatment of trans women. For this purpose, it provides an overview of the physiological action of progesterone in target tissues and speculates on possible benefits for gender transitioning. Progesterone is expected to exert moderate anti-androgen action through suppression of the hypothalamic-pituitary-gonadal axis and inhibition of the conversion of testosterone to dihydrotestosterone. It may also contribute to breast maturation. In the long-term, progesterone could prevent bone loss and protect cardiovascular health. The potential benefits are mainly inferred by extrapolating evidence from biological actions in cisgender women and medical assumptions and hence, clinicians need to be cautious when applying these data into practice. Further research is needed to ascertain the efficacy and safety of progesterone in current hormonal regimens.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10558402/

Record 674

pmcid: PMC9438014

pmid: 36041762

doi: 10.1136/bmjopen-2022-062478

title: Protocol for an observational study investigating hormones triggering the onset of sustained lactation: the INSIGHT study.

authors

- Rostom H
- Meng X
- Price H
- Fry A
- Elajnaf T
- Humphrey R
- Guha N
- James T
- Kennedy SH
- Hannan FM

journal: BMJ open

journalAbbr: BMJ Open

year: 2022

pubdate: 2022 Aug 30

volume: 12

issue: 8

pages: e062478

abstract: INTRODUCTION: Lactation is a hormonally controlled process that promotes infant growth and neurodevelopment and reduces the long-term maternal risk of diabetes, cardiovascular disease and breast cancer. Hormones, such as prolactin and progesterone, mediate mammary development during pregnancy and are critical for initiating copious milk secretion within 24-72 hours post partum. However, the hormone concentrations mediating lactation onset are ill defined. METHODS AND ANALYSIS: The primary objective of the investigating hormones triggering the onset of sustained lactation study is to establish reference intervals for the circulating hormone concentrations initiating postpartum milk secretion. The study will also assess how maternal factors such as parity, pregnancy comorbidities and complications during labour and delivery, which are known to delay lactation, may affect hormone concentrations. This single-centre observational study will recruit up to 1068 pregnant women over a 3-year period. A baseline blood sample will be obtained at 36 weeks' gestation. Participants will be monitored during postpartum days 1-4. Lactation onset will be reported using a validated breast fullness scale. Blood samples will be collected before and after a breastfeed on up to two occasions per day during postpartum days 1-4. Colostrum, milk and spot urine samples will be obtained on a single occasion. Serum hormone reference intervals will be calculated as mean±1.96 SD, with 90% CIs determined for the upper and lower reference limits. Differences in hormone values between healthy breastfeeding women and those at risk of delayed onset of lactation will be assessed by repeated measures two-way analysis of variance or a mixed linear model. Correlations between serum hormone concentrations and milk composition and volume will provide insights into the endocrine regulation of milk synthesis. ETHICS AND DISSEMINATION: Approval for this study had been granted by the East of England-Cambridgeshire and Hertfordshire Research Ethics Committee (REC No. 20/EE/0172), by the Health Research Authority (HRA), and by the Oxford University Hospitals National Health Service Foundation Trust. The findings will be published in high-ranking journals and presented at national and international conferences. TRIAL REGISTRATION NUMBER: ISRCTN12667795.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9438014/

Record 675

pmcid: PMC8897593

pmid: 35212265

doi: 10.1530/EDM-21-0100

title: Pseudo-pheochromocytoma due to obstructive sleep apnea: a case report.

authors

- Jazdarehee A
- Huget-Penner S
- Pawlowska M

journal: Endocrinology, diabetes & metabolism case reports

journalAbbr: Endocrinol Diabetes Metab Case Rep

year: 2022

pubdate: 2022 Jan 1

volume: 2022

issue:

pages:

abstract: SUMMARY: Obstructive sleep apnea (OSA) is a condition of intermittent nocturnal upper airway obstruction. OSA increases sympathetic drive which may result in clinical and biochemical features suggestive of pheochromocytoma. We present the case of a 65-year-old male with a 2.9-cm left adrenal incidentaloma on CT, hypertension, symptoms of headache, anxiety and diaphoresis, and persistently elevated 24-h urine norepinephrine (initially 818 nmol/day (89-470)) and normetanephrine (initially 11.2 µmol/day (0.6-2.7)). He was started on prazosin and underwent left adrenalectomy. Pathology revealed an adrenal corticoadenoma with no evidence of pheochromocytoma. Over the next 2 years, urine norepinephrine and normetanephrine remained significantly elevated with no MIBG avid disease. Years later, he was diagnosed with severe OSA and treated with continuous positive airway pressure. Urine testing done once OSA was well controlled revealed complete normalization of urine norepinephrine and normetanephrine with substantial symptom improvement. It was concluded that the patient never had a pheochromocytoma but rather an adrenal adenoma with biochemistry and symptoms suggestive of pheochromocytoma due to untreated severe OSA. Pseudo-pheochromocytoma is a rare presentation of OSA and should be considered on the differential of elevated urine catecholamines and metanephrines in the right clinical setting. LEARNING POINTS: Obstructive sleep apnea (OSA) is a common condition among adults. OSA may rarely present as pseudo-pheochromocytoma with symptoms of pallor, palpitations, perspiration, headache, or anxiety. OSA should be considered on the differential of elevated urine catecholamines and metanephrines, especially in patients with negative metaiodobenzylguanidine (MIBG) scan results.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8897593/

Record 676

pmcid: PMC11092986

pmid: 38751524

doi: 10.21037/tbcr-21-37

title: Pseudoangiomatous stromal hyperplasia causing massive unilateral breast enlargement-a case report.

authors

- Brodie A
- Eltatawy A
- Lambert K

journal: Translational breast cancer research : a journal focusing on translational research in breast cancer

journalAbbr: Transl Breast Cancer Res

year: 2022

pubdate: 2022

volume: 3

issue:

pages: 19

abstract: BACKGROUND: Pseudoangiomatous stromal hyperplasia (PASH) is an uncommon mesenchymal benign breast neoplasm. PASH may present as a rapidly growing mass in pre-menopausal women. It is important to rule out other causes of a breast lump including fibroadenoma, hamartoma, phyllodes tumour and invasive adenocarcinoma. To our knowledge this is the largest case of PASH tumour in the English literature and the first to be managed with mastectomy and immediate breast implant reconstruction. CASE DESCRIPTION: A 45-year-old woman with a history of bilateral breast augmentation for cosmesis and no other co-morbidities, presented with gross asymmetry between her right and left breasts on 13 th December 2020. She complained about pain and the noticeable difference in clothing. She underwent triple assessment (history and examination, imaging, and biopsy). Bilateral mammograms showed the right breast had a possible mass. On ultrasound imaging there was evidence of extensive ill-defined hypoechoic changes. Core biopsies and a vacuum assisted core biopsy were taken from the mass and came back as PASH/fibrocystic change on 12 th

January 2021. Surgical options were discussed, and after some consideration she decided on mastectomy and immediate implant reconstruction on 15 th March 2021. She underwent the operation on 9 th September 2021 and the specimen weight was 1.911 kg and measured 27 cm × 22.6 cm × 7.4 cm which is the largest reported PASH tumour. She made an uncomplicated recovery and was seen in clinic 4 weeks later and was happy with the cosmetic outcome. PASH can present as incidental microscopic foci in biopsies to diffuse enlargement of the breast. PASH has no specific radiological or diagnostic features, therefore is diagnosed based on histology. PASH is not associated with an increased risk of malignancy and the management is dependent on the presentation. When PASH is discovered incidentally or is asymptomatic it can be followed up with serial mammography to assess growth. If there are any suspicious features or discordance following triple assessment or symptoms such as discomfort surgical excision is indicated. CONCLUSIONS: PASH should be considered as part of the differential diagnosis for rapid breast enlargement. Treatment options should be discussed early with the patient to prevent progression. For rapid breast enlargement mastectomy and immediate reconstruction should be considered and discussed with the patient.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11092986/

Record 677

pmcid: PMC9218285

pmid: 35755115

doi: 10.1016/j.radcr.2022.05.033

title: Pseudoangiomatous stromal hyperplasia in a healthy young adult male.

authors

- Owji SC
- Shroff N
- Robinson AS
- Posleman Monetto FE

journal: Radiology case reports

journalAbbr: Radiol Case Rep

year: 2022

pubdate: 2022 Sep

volume: 17

issue: 9

pages: 2919-2922

abstract: This case report describes the occurrence of a rapidly enlarging pseudoangiomatous stromal hyperplasia (PASH) tumor in a 20-year-old male patient. The diagnosis was made via tomosynthesis and ultrasound-guided biopsy with pathological correlation consistent with PASH. The patient's case was discussed, and he was recommended to undergo surgical resection of the mass to alleviate symptoms due to its large size. Surgical pathology confirmed the original diagnosis and the patient had an uncomplicated postoperative course. Here, we exhibit our imaging findings; review classic presentations of PASH on mammography, ultrasound, and MRI; and discuss histological characteristics of this benign entity.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9218285/

Record 678

pmcid: PMC9361800

pmid: 35957783

doi: 10.1002/ccr3.6221

title: Pseudoangiomatous stromal hyperplasia of the breast: A case report.

authors

- Tahmasebi S
- Khorsand Rahim Zadeh A
- Zangouri V
- Akrami M
- Ghoddusi Johari M
- Talei A

journal: Clinical case reports

journalAbbr: Clin Case Rep

year: 2022

pubdate: 2022 Aug

volume: 10

issue: 8

pages: e6221

abstract: Pseudoangiomatous stromal hyperplasia (PASH) is a rare benign breast entity scarcely reported in the medical literature. Its pathogenesis, etiology, and optimal treatment are still unknown. PASH tumors have a broad spectrum of clinical presentations and might be mistaken for malignancies. The authors present six patients diagnosed with PASH.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9361800/

Record 679

pmcid: PMC9710853

pmid: 36465764

doi: 10.7759/cureus.30925

title: Quality of Life Assessment for Men With Gynecomastia in Saudi Arabia.

authors

- Alnaim MF
- Alraihan JI
- Al Rabiah NM
- Zogel B
- Alfaifi SM
- Azam AN
- Baghdadi AO
- Alhusaini BK

journal: Cureus

journalAbbr: Cureus

year: 2022

pubdate: 2022 Oct

volume: 14

issue: 10

pages: e30925

abstract: BACKGROUND: Gynecomastia, defined as a benign proliferation of the breasts in males, is a common pathology of breasts among adolescent males. The root cause behind the condition is the hormonal imbalance between androgens and estrogens at the time of puberty. Different treatment options can be used; however, surgery is the preferred option. The quality of life (QoL) of affected Individuals is usually affected in all aspects. OBJECTIVES: The aim of the study is to assess the QoL of male Saudi patients diagnosed with gynecomastia and their desire to undergo surgical treatment. METHODS: A cross-sectional study was conducted using an Arabic self-administered online questionnaire that targeted males in Saudi Arabia and was distributed throughout the kingdom. RESULTS: A total of 681 participants were involved in this study. Most of the participants were Saudi (n = 607; 90.6%) and married (n = 158; 79%). Approximately, half of the participants were between 18 and 29 years of age (n = 337; 49.5%), while 41.1% and 9.3% were between 30 and 49 years and more than 50 years old, respectively. About 29.5% of the participants were from the middle region, while 26.7% of them were from the northern region, followed by 19.5% of participants from the western region, and only 14.4% and 9.8% from the eastern or southern region. On analysis of different domains, there was no statistically significant difference between participants with gynecomastia and the control group in QoL. CONCLUSIONS: Patients showed no statistically significant change in the QoL between those diagnosed with gynecomastia and those in the control group. Also, more than one-third of our patients did not want to undergo breast reduction surgery.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9710853/

Record 680

pmcid: PMC9827728

pmid: 35581122

doi: 10.1016/j.urolonc.2022.04.009

title: Quality of life in low-income men after surgical castration for metastatic prostate cancer.

authors

- Gaither TW
- Kwan L
- Villatoro J
- Litwin MS

journal: Urologic oncology

journalAbbr: Urol Oncol

year: 2022

pubdate: 2022 Jul

volume: 40

issue: 7

pages: 343.e7-343.e14

abstract: OBJECTIVE: To compare health-related quality of life in men who underwent surgical vs. medical castration for metastatic prostate cancer. METHODS: We analyzed data from a prospective cohort of men enrolled in a statewide public health program that provides care for prostate cancer among low-income, uninsured men from 2001 to 2020. Outcome measures included the RAND SF-12 and the UCLA Prostate Cancer Index (PCI) at baseline and every 6 months. We used generalized estimating equations to assess the independent impact of surgical vs. medical castration on health-related quality of life. RESULTS: Among men with metastatic prostate cancer, 27 underwent orchiectomy, and 274 underwent medical castration. Median cohort age at enrollment was 61.3 years (IQR 56-65); 239 (79%) men had less than a high school education. Average follow-up was 8 months (range 0-45) since study enrollment. Seventy percent of patients within the surgical castration group had their orchiectomy prior to study enrollment (median months since orchiectomy at study enrollment was 9 months, IQR 1-43). Similarly, 59% of patients within the medical castration group had begun ADT prior to study enrollment (median months since ADT initiation at study enrollment was 4 months,

IQR 1-12). The majority (66%) had metastatic disease at diagnosis. The 2 groups did not differ in age, race/ethnicity, education, monthly income, baseline PSA, Gleason score, or percent metastatic at diagnosis. SF-12 domains did not differ between those who underwent surgical vs. medical castration (on average throughout follow-up, physical component difference -2.0, 95% CI -8.0-3.9 and mental component difference -1.0, 95% CI -5.4-+3.4). Patients treated with orchiectomy reported better urinary function than those who underwent medical castration (+16 point, 95%CI 5.3-26). CONCLUSIONS: Surgical castration did not negatively impact general or disease-specific quality of life. The finding of improved urination after orchiectomy merits further inquiry. This may inform urologists' discussion of surgical vs. medical options for men with castration-sensitive metastatic prostate cancer.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9827728/

Record 681

pmcid: PMC9167639

pmid: 35677738

doi: 10.7759/cureus.25666

title: Radio Electric Asymmetric Conveyer Tissue Reparative Treatment on Post-surgical Breast Skin Necrosis. A Report of Four Cases.

authors

- Fontani V
- Coelho Pereira JA
- Rinaldi S

journal: Cureus

journalAbbr: Cureus

year: 2022

pubdate: 2022 Jun

volume: 14

issue: 6

pages: e25666

abstract: Breast surgical treatments for both tumors and aesthetic reasons are very frequent. The nipple-areola complex (NAC) ischemia is a possible complication after breast surgery. This lesion can be devastating for the patient in the post-surgical course and can lead to final epidermolysis. The necrosis is generally attributed to vascular compromise or excessive tension of the flaps. Actually, the phenomena that prevent spontaneous repair are due to variations in the endogenous electrical potential at the cellular level. In damaged tissues, the electric potential difference across the epithelium is often profoundly altered. In this manuscript, we are presenting four cases of NAC necrosis that were successfully treated with reparative tissue optimization (TO-RPR) treatment of the Radio Electric Asymmetric Conveyer (REAC) technology. REAC technology was conceived to overcome the limits of exogenous electrical stimulations. Instead of administering an electrical stimulus that imposes itself on the endogenous bioelectric activity (EBA), the REAC technology restores the correct potential difference inside the tissues, which is essential for all reparative and regenerative processes. The REAC treatment applied was able to promote a fast-healing process of the necrosis of the NAC following surgery of the breast.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9167639/

Record 682

pmcid: PMC9238198

pmid: 36237367

doi: 10.3348/jksr.2021.0011

title: Radiologic Findings of Polyacrylamide Gel Mammoplasty and Its Complications: A Report of Four Case Series and Review of Literature.

authors

- Kim JH
- Hwang JY
- Moon JH
- Yang I
- Woo JY

journal: Taehan Yongsang Uihakhoe chi

journalAbbr: Taehan Yongsang Uihakhoe Chi

year: 2022

pubdate: 2022 Jan

volume: 83

issue: 1

pages: 230-238

abstract: Polyacrylamide gel (PAAG) has been used for augmentation mammoplasty in certain countries, such as China or Eastern European countries. However, owing to various complications, its use is currently banned in most countries. Nevertheless, because of the migratory stream of female from these countries, we also encounter patients with PAAG mammoplasty in Korea. Owing to the various imaging features of PAAG mammoplasty, it may interfere with the identification of malignancy or other pathologic conditions. Therefore, it is important to identify the imaging findings related to PAAG mammoplasty. However, there is limited literature on the radiologic findings of complications related to PAAG mammoplasty in Korea. Herein we report four cases of PAAG mammoplasty, the complications faced, and depict various radiologic features through multimodality imaging.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9238198/

Record 683

pmcid: PMC8670371

pmid: 34779392

doi: 10.5414/CP204034

title: Randomized, double-blind, two-way crossover bioequivalence and adhesion study, in healthy women, of a transdermal contraceptive patch with a newly sourced adhesive component at the end of shelf life vs. the EVRA patch at the beginning of shelf life.

authors

- Sanga M
- Vaughan S
- Nangosyah J
- Scholz V
- Fonseca S

journal: International journal of clinical pharmacology and therapeutics

journalAbbr: Int J Clin Pharmacol Ther

year: 2022

pubdate: 2022 Jan

volume: 60

issue: 1

pages: 67-78

abstract: OBJECTIVE: Evaluate bioequivalence, based on norelgestromin (NGMN) and ethinyl estradiol (EE) plasma concentrations, and adhesion of a transdermal contraceptive patch containing a newly sourced adhesive component (test) at end of shelf life (EOSL) vs. the marketed EVRA patch (reference) at beginning of shelf life (BOSL). MATERIALS AND METHODS: In this randomized, double-blind, two-way crossover study, healthy women received a single, 7-day application of test and reference patches in 4 sequences: two 11-day treatment periods separated by a 21-day washout. Assessments included NGMN and EE pharmacokinetics (PK), adhesion (per European Medicines Agency (EMA) 5-point scale), irritation potential and application-site reactions, and tolerability. Patches were bioequivalent if 90% CIs of geometric mean ratios (GMRs) of test/reference for C max , AUC 168h , AUC 0-tlast , and AUC ∞ were 80 - 125%. Patch adhesion was comparable if ratios of geometric mean cumulative adhesion percentages were ≥ 90%. RESULTS: 68 women were randomized, and 62 completed both treatments. 55 and 59 participants in the reference and test group, respectively, had patch adhesion ≥ 80% (EMA score 0 - 1) at end of treatment. Bioequivalence was demonstrated: GMRs for pharmacokinetic (PK) parameters ranged from 102.76 - 105.57% for NGMN and 93.78 - 94.80% for EE, and associated 90% CIs were fully within the bioequivalence acceptance range (80 - 125%) for both. The patches had comparable adhesion properties (GMR, 101.4% (90% CI: 99.2 - 103.6)) and incidences of treatment-emergent adverse events. CONCLUSION: NGMN-EE transdermal test patch at EOSL was bioequivalent to the marketed patch at BOSL, supporting widening the product's shelf-life specification. Adhesive properties and safety profiles were comparable between patches.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8670371/

Record 684

pmcid: PMC9812521

pmid: 36619589

doi: 10.3389/fgwh.2022.1012463

title: Recent advances in understanding adverse effects associated with drugs targeting the serotonin receptor, 5-HT GPCR.

authors

- Jamu IM
- Okamoto H

journal: Frontiers in global women's health

journalAbbr: Front Glob Womens Health

year: 2022

pubdate: 2022

volume: 3

issue:

pages: 1012463

abstract: It has been acknowledged that more women suffer from adverse effects of drugs than men globally. A group of drugs targeting serotonin [5-hydroxytryptamine] (5-HT) binding G-protein-coupled receptors (GPCRs) have been reported to preferentially affect women more than men, causing adverse effects such as breast cancer and infertility. 5-HT GPCR-targeted drugs in the central nervous system (CNS) manage psychiatric conditions, such as depression or bipolar and in the peripheral nervous system (PNS) treat migraines. Physiological characteristics such as specific types of hormones, higher body fat density and smaller body mass in women result in disparities in pharmacodynamics of drugs, thus explaining sex-related differences in the observed adverse effects. In this review, we discuss the side effects of drugs targeting 5-HT GPCRs based on serotonin's roles in the CNS and PNS. We have systematically reviewed adverse effects of drugs targeting 5-HT

GPCR using information from the Food and Drug Administration and European Medicines Agency. Further information on drug side effects and receptor targets was acquired from the SIDER and DrugBank databases, respectively. These drugs bind to 5-HT GPCRs in the CNS, namely the brain, and PNS such as breasts, ovaries and testes, potentially causing side effects within these areas. Oestrogen affects both the biosynthesis of 5-HT and the densities of 5-HT GPCRs in given tissues and cells. 5-HT GPCR-targeting drugs perturb this process. This is likely a reason why women are experiencing more adverse effects than men due to their periodic increase and the relatively high concentrations of oestrogen in women and, thus a greater incidence of the oestrogen-mediated 5-HT system interference. In addition, women have a lower concentration of serotonin relative to men and also have a relatively faster rate of serotonin metabolism which might be contributing to the former. We discuss potential approaches that could mitigate at least some of the adverse effects experienced by women taking the 5-HT GPCR-targeting drugs.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9812521/

Record 685

pmcid: PMC9629063

pmid: 36308406

doi: 10.1080/07853890.2022.2138961

title: Recognizing skin conditions in patients with cirrhosis: a narrative review.

authors

- Liu Y
- Zhao Y
- Gao X
- Liu J
- Ji F
- Hsu YC
- Li Z
- Nguyen MH

journal: Annals of medicine

journalAbbr: Ann Med

year: 2022

pubdate: 2022 Dec

volume: 54

issue: 1

pages: 3017-3029

abstract: Background: The skin is a major target organ for extrahepatic manifestations of liver diseases, and dermatologic abnormalities are common in patients with hepatic disorders. Clinical examination of the skin, nails and hair can allow for appropriate recognition, early diagnosis and treatment of liver diseases, and improvement in the quality of life and life expectancy of affected patients. Methods: We searched 3 databases (Pubmed,Medline and Embase) and selected studies about cirrhosis related skin manifestations and their pathophysiology. Results: A total of 73 articles were included in the review. Studies displayed the spectrum of cutaneous manifestations related to hormonal and vascular changes as well as nail and hair changes in patients with cirrhosis and/or portal hypertension. Conclusion: Cutaneous alterations are important clues or potential indications in the diagnosis of liver cirrhosis. Familiarity with skin conditions can be promptly diagnosed and appropriate management initiated.KEY MESSAGESManifestations of the liver and skin disorders are interrelated in various ways. Cutaneous changes may be the first clue that a patient has liver disease.The skin is a major target organ for extrahepatic manifestations of liver diseases. A broad range of cutaneous alterations can be present in patients with cirrhosis, such as vascular, nail, hair, hormonal changes, etc.Recognizing these

signs is crucial so that potential underlying diseases including liver disease can be promptly diagnosed and appropriate management timely initiated.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9629063/

Record 686

pmcid: PMC9469751

pmid: 36120227

doi: 10.7759/cureus.27991

title: Reversible Gynecomastia in HIV-Infected Man Treated With Triple Antiretroviral Therapy Containing Efavirenz: A Case Report.

authors

- Essafi MA
- Elazizi L
- Aynaou H
- Salhi H
- El Ouahabi H

journal: Cureus

journalAbbr: Cureus

year: 2022

pubdate: 2022 Aug

volume: 14

issue: 8

pages: e27991

abstract: Gynecomastia is benign hypertrophy of male breast glandular tissue, either unilateral or bilateral, secondary to increased estrogen/testosterone ratio (elevated estrogen level, decreased testosterone levels, or both). The condition can be related to a medical disease or caused by some drugs. Since the introduction of triple antiretroviral therapy (TAT), we have seen an improvement in the prognosis of human immunodeficiency virus (HIV) infection. Here we report the case of a 53-year-old man receiving follow-up care in Internal Medicine for HIV infection receiving TAT (tenofovir/efavirenz/emtricitabine). After one year, the patient presented in the Department of Endocrinology, Diabetology, Metabolic Diseases, and Nutrition of Hassan II University Hospital Center, Fez, with bilateral gynecomastia. Hormonal exploration did not reveal any abnormality, so the gynecomastia was attributed to efavirenz use. The regimen was replaced by tenofovir, lamivudine, and dolutegravir. The gynecomastia was resolved within two months of discontinuing efavirenz. In summary, we think that secondary gynecomastia should be suspected and screened in HIV patients receiving efavirenz-containing regimens.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9469751/

Record 687

pmcid: PMC9187173

pmid: 35702542

doi: 10.1097/GOX.0000000000004355

title: Review of Tuberous Breast Deformity: Developments over the Last 20 Years.

authors

- Lozito A

- Vinci V
- Talerico E
- Asselta R
- Di Tommaso L
- Agnelli B
- Klinger M
- Klinger F

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2022

pubdate: 2022 May

volume: 10

issue: 5

pages: e4355

abstract: UNLABELLED: Tuberous breast (TB) deformity is a condition characterized by alterations in breast morphology and tissue structure with high prevalence in the general population. The literature provides sparse descriptions of TB, as not many investigations on the condition have been conducted. The aim of this review was to analyze and provide a holistic overview on the morphological characteristics of the TB. METHODS: A review of current literature was performed using the PubMed database from 2001 to 2021. The key words used for the review included "tuberous breast," "constricted breast," and "stenotic breast." We included articles that analyzed the anatomic and histologic characteristics of TB. RESULTS: From 213 articles, only 42 met the inclusion criteria. A total of 171 articles were excluded, as they were letters, not related to the condition, or were written in a foreign language. The studies in this review drew on hypothesis on the embryological origin of TB and analyzed the composition of TB tissues, consisting in a constricting fibrous ring, made of longitudinally arranged collagen and elastic fibers. Furthermore, the review reports the different anatomical and surgical classifications, as well as the various surgical corrective procedures developed throughout history up to 2021. CONCLUSION: The review describes all etiological, epidemiologic, anatomical, histological, and surgical characteristics of tuberous breast.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9187173/

Record 688

pmcid: PMC9121093

pmid: 35599770

doi: 10.3389/fpsyt.2022.874705

title: Risperidone Induced Hyperprolactinemia: From Basic to Clinical Studies.

authors

- Stojkovic M
- Radmanovic B
- Jovanovic M
- Janjic V
- Muric N
- Ristic DI

journal: Frontiers in psychiatry

journalAbbr: Front Psychiatry

year: 2022

pubdate: 2022

volume: 13

issue:

pages: 874705

abstract: Risperidone is one of the most commonly used antipsychotics (AP), due to its safety and efficacy in reducing psychotic symptoms. Despite the favorable side effect profile, the therapy is accompanied by side effects due to the non-selectivity of this medicine. This review will briefly highlight the most important basic and clinical findings in this area, consider the clinical effects of AP-induced hyperprolactinemia (HPL), and suggest different approaches to the treatment.The route of application of this drug primarily affects the daily variation and the total concentration of drug levels in the blood, which consequently affects the appearance of side effects, either worsening or even reducing them. Our attention has been drawn to HPL, a frequent but neglected adverse effect observed in cases treated with Risperidone and its secondary manifestations. An increase in prolactin levels above the reference values result in impairment of other somatic functions (lactation, irregular menses, fertility) as well as a significant reduction in quality of life. It has been frequently shown that the side effects of the Risperidone are the most common cause of non-compliance with therapy, resulting in worsening of psychiatric symptoms and hospitalization. However, the mechanism of Risperidone-induced HPL is complicated and still far from fully understood. Most of the preclinical and clinical studies described in this study show that hyperprolactinemia is one of the most common if not the leading side effect of Risperidone therefore to improve the quality of life of these patients, clinicians must recognize and treat HPL associated with the use of these drugs.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9121093/

Record 689

pmcid: PMC9221541

pmid: 35740965

doi: 10.3390/biom12060840

title: Role of Seipin in Human Diseases and Experimental Animal Models.

authors

- Li Y
- Yang X
- Peng L
- Xia Q
- Zhang Y
- Huang W
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- Jia D

journal: Biomolecules

journalAbbr: Biomolecules

year: 2022

pubdate: 2022 Jun 17

volume: 12

issue: 6

pages:

abstract: Seipin, a protein encoded by the Berardinelli-Seip congenital lipodystrophy type 2 (BSCL2) gene, is famous for its key role in the biogenesis of lipid droplets and type 2 congenital generalised lipodystrophy

(CGL2). BSCL2 gene mutations result in genetic diseases including CGL2, progressive encephalopathy with or without lipodystrophy (also called Celia's encephalopathy), and BSCL2 -associated motor neuron diseases. Abnormal expression of seipin has also been found in hepatic steatosis, neurodegenerative diseases, glioblastoma stroke, cardiac hypertrophy, and other diseases. In the current study, we comprehensively summarise phenotypes, underlying mechanisms, and treatment of human diseases caused by BSCL2 gene mutations, paralleled by animal studies including systemic or specific Bscl2 gene knockout, or Bscl2 gene overexpression. In various animal models representing diseases that are not related to Bscl2 mutations, differential expression patterns and functional roles of seipin are also described. Furthermore, we highlight the potential therapeutic approaches by targeting seipin or its upstream and downstream signalling pathways. Taken together, restoring adipose tissue function and targeting seipin-related pathways are effective strategies for CGL2 treatment. Meanwhile, seipin-related pathways are also considered to have potential therapeutic value in diseases that are not caused by BSCL2 gene mutations.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9221541/

Record 690

pmcid: PMC9810484

pmid: 36620820

doi: 10.7759/cureus.32184

title: Subclavian Vein Stenosis Imitating Inflammatory Breast Cancer.

authors

- Judd AC
- Weinkauf C
- Erdrich J

journal: Cureus

journalAbbr: Cureus

year: 2022

pubdate: 2022 Dec

volume: 14

issue: 12

pages: e32184

abstract: Unilateral breast erythema, edema, and peau d'orange are classically associated with inflammatory breast cancer. However, occasionally this constellation of symptoms is seen with other causes. Maintaining a broad differential can therefore save a prospective patient from months of worry about a possible cancer diagnosis, untreated symptoms, and unnecessary and expensive tests. Here we present the case of a 75-year-old woman with a history of pacemaker placement complicated by left upper extremity deep venous thrombosis (DVT) who subsequently developed left breast peau d'orange , swelling, and erythema. After initially being worked up for inflammatory breast cancer, including multiple breast biopsies, she was then referred to specialists in cardiology, allergy, pulmonology, rheumatology, dermatology, lymphedema therapy, and vascular surgery undergoing an exhaustive workup that spanned nearly a year. Eventually, a venogram was performed, which revealed complete occlusion of her left subclavian vein. After undergoing angioplasty and stenting, her symptoms resolved.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9810484/

Record 691

pmcid: PMC8815117

pmid: 35115037

doi: 10.1186/s40001-021-00628-1

title: Successful ECMO-assisted open chest cardiopulmonary resuscitation in a postpartum patient with delayed amniotic fluid embolism.

authors

- Wu Y
- Luo J
- Chen T
- Zhan H
- Liu J
- Chen J
- Wang S

journal: European journal of medical research

journalAbbr: Eur J Med Res

year: 2022

pubdate: 2022 Feb 3

volume: 27

issue: 1

pages: 19

abstract: BACKGROUND: Amniotic fluid embolism (AFE) is a rare but potentially dangerous severe obstetrics complication, which is accompanied by an incidence between 1.9 and 6.1 per 100,000 births. CASE PRESENTATION: Here, we report an AFE case after cesarean delivery diagnosed on a cardiac arrest complicated by acute respiratory distress syndrome and coagulopathy. Diagnosis, risk factors and pathophysiology for AFE have been fully discussed, besides, extracorporeal membrane oxygenation in the early management of cardiac arrest was used, describing the indication, efficacy and successful performed of open-chest cardiopulmonary resuscitation for the patient. CONCLUSION: In AFE with cute cardiovascular collapse, extracorporeal membrane oxygenation support can be considered as the alternative therapies.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8815117/

Record 692

pmcid: PMC9411179

pmid: 36035230

doi: 10.1016/j.jvscit.2022.04.018

title: Superior vena cava syndrome as unilateral right breast enlargement.

authors

- Khera SY

journal: Journal of vascular surgery cases and innovative techniques

journalAbbr: J Vasc Surg Cases Innov Tech

year: 2022

pubdate: 2022 Sep

volume: 8

issue: 3

pages: 477-479

abstract: This case report describes a postmenopausal woman presenting to the breast surgery clinic with right breast enlargement, new spider veins on the chest, dyspnea, and facial swelling. She was treated for lung

cancer. Imaging showed her right hilar mass causing critical superior vena cava (SVC) stenosis. She was transferred to thoracic surgery, underwent SVC stent placement, and her symptoms improved. SVC syndrome as a cause of right breast enlargement is unusual; therefore, carefully reviewing symptoms, medical history, and physical examination is crucial for diagnosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9411179/

Record 693

pmcid: PMC9451573

pmid: 36128591

doi: 10.14740/wjon1513

title: Syringomatous Tumor of the Nipple.

authors

- Favre NM
- Sabih Q
- L'Huillier JC
- Takabe K
- Cappuccino H

journal: World journal of oncology

journalAbbr: World J Oncol

year: 2022

pubdate: 2022 Aug

volume: 13

issue: 4

pages: 235-240

abstract: Syringomatous tumor of the nipple (SyT), previously known as syringomatous adenoma of the nipple (SAN) was originally described in 1983 as a rare benign tumor of the sweat duct. It is a rare benign tumor arising from the skin adnexal eccrine glands that can be misdiagnosed as invasive carcinoma due to its infiltrative nature, resulting in overtreatment. We report clinical, histopathologic, and surgical findings of a case of infiltrating syringomatous adenoma in a 40-year-old female as well as a literature review of previous cases. A high index of suspicion is required to make the diagnosis of SyT and prevent overtreatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9451573/

Record 694

pmcid: PMC9091814

pmid: 35574545

doi: 10.3389/fsurg.2022.865379

title: The Application of Anatomy Combined With Ultrasound Knife in Transaxillary Endoscopic Biplane Breast Augmentation.

authors

- Xiong J
- Hou Q
- Hu Z
- Gao Y

- Lu L
- Sun M
- Hu H
- Qian Y
- Wang H
- Jiang H

journal: Frontiers in surgery

journalAbbr: Front Surg

year: 2022

pubdate: 2022

volume: 9

issue:

pages: 865379

abstract: OBJECTIVE: We aim to clarify the vascular and nerve anatomy of the breast and combine it with an ultrasound knife to use in transaxillary endoscopic biplane breast augmentation. METHODS: This study is a retrospective review of patients undergoing transaxillary endoscopic biplane breast augmentation between October and October 2021. Related variables were collected using a standardized data collection template. The detailed process of the transaxillary endoscopic biplane breast augmentation under anatomy instruction is carefully described in this study, and the postoperative effect was closely observed. RESULTS: Sixty-three female patients underwent transaxillary endoscopic biplane breast augmentation. The average implants volume counted 242.46 ± 31.34 cc, and the average operation time was 155.92 ± 22.34 min. Patients were followed up for a mean of 13.67 months (range, 3-27 months), and most of the patients achieved good postoperative results and no severe complications and were satisfied with both appearance and function. CONCLUSIONS: The application of anatomy combined with an ultrasound knife in transaxillary endoscopic biplane breast augmentation is a promising way to achieve good breast shapes with high patient satisfaction and is worthy of clinical promotion and application.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9091814/

Record 695

pmcid: PMC9805125

pmid: 35833603

doi: 10.1002/pon.6001

title: The impact of hegemonic masculine ideals on self-esteem in prostate cancer patients undergoing androgen deprivation therapy (ADT) compared to ADT-naïve patients.

authors

- Gentili C
- McClean S
- McGeagh L
- Bahl A
- Persad R
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journal: Psycho-oncology

journalAbbr: Psychooncology

year: 2022

pubdate: 2022 Nov

volume: 31

issue: 11

pages: 1958-1971

abstract: PURPOSE: Androgen deprivation therapy (ADT) for Prostate Cancer (PCa) is associated with side effects that could lead to negative body image and low masculine self-esteem of survivors. We compared a group of PCa survivors following ADT with ADT-naïve patients, expecting the ADT group to show lower masculine self-esteem. We also expected patients with hegemonic masculinity ideals to show poorer masculine self-esteem and we hypothesized that ADT would moderate this relationship, expecting PCa patients on ADT with stronger hegemonic ideals to show the worst masculine self-esteem scores among study participants. METHODS: We compared 57 PCa survivors on ADT (M age = 64.16 (7.11)) to 59 ADT-naïve patients (M age = 65.25 (5.50)), on the Masculine Self-Esteem Scale (MSES), Body Image Scale (BIS), and Hegemonic Masculinity Ideals Scale (HMIS). RESULTS: While the two groups did not significantly differ on masculine self-esteem ($F [1, 115] = 3.46$, $p = 0.065$, $\eta p 2 = 0.029$) and body image ($F [1, 115] = 3.46$, $p = 0.065$, $\eta p 2 = 0.029$), younger age was significantly associated with higher body image issues ($F [1, 115] = 8.63$, $p < 0.01$, $\eta p 2 = 0.071$, $\beta = -0.30$). Hegemonic masculinity significantly predicted more masculine self-esteem related issues ($t (2, 114) = 2.31$, $\beta = 0.375$, $p < 0.05$). ADT did not moderate this relationship. CONCLUSIONS: The results suggest that endorsing hegemonic masculinity could represent a risk factor for low masculine self-esteem regardless of ADT status and that younger age is associated with negative body image among PCa survivors. IMPLICATIONS: These results suggest the importance of inclusion of topics related to hegemonic masculinity when providing support to PCa survivors, both when discussing treatment side effects, as well as in the later phases of survivorship. This pilot also suggests that younger PCa survivors might benefit from body-image focused support regardless of treatment plan.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9805125/

Record 696

pmcid: PMC9736970

pmid: 36499573

doi: 10.3390/ijms232315250

title: The Role of Hormones in Hidradenitis Suppurativa: A Systematic Review.

authors

- Abu Rached N
- Gambichler T
- Dietrich JW
- Ocker L
- Seifert C
- Stockfleth E
- Bechara FG

journal: International journal of molecular sciences

journalAbbr: Int J Mol Sci

year: 2022

pubdate: 2022 Dec 3

volume: 23

issue: 23

pages:

abstract: Hidradenitis suppurativa (HS) is a chronic inflammatory disease manifesting in inverse body regions. In a systematic review, the role of hormones in HS will be presented to better understand the pathomechanisms of HS. The review is based on the PRISMA criteria. Systematic research was carried out using keywords. Subsequently, the data were analyzed based on the clinical response and other relevant information. The main focus of our systematic review was on HS manifestation, exacerbation, sex hormones, antiandrogen therapy, thyroid function, polycystic ovary syndrome, insulin resistance, and adipokines. In HS, there appears to be a dysregulated adipokine release that is shifted towards pro-inflammatory adipokines. Insulin resistance is significantly more common in HS than in healthy patients regardless of BMI, age, and gender. Insulin resistance in HS patients leads to further cardiovascular disease. The mechanism of insulin resistance and role of adipokines should be investigated in future studies to better provide the pathomechanisms of HS. The role of androgens seems to be important in a certain subgroup of female patients. Anti-androgenic therapy can be useful and helpful in some patients. However, further studies are needed to better understand the hormonal relationship in HS.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9736970/

Record 697

pmcid: PMC9266871

pmid: 35832720

doi: 10.5114/aoms.2019.88280

title: The role of oestrogen and progesterone receptors in gigantomastia.

authors

- Kasielska-Trojan A
- Danilewicz M
- Strużyna J
- Bugaj M
- Antoszewski B

journal: Archives of medical science : AMS

journalAbbr: Arch Med Sci

year: 2022

pubdate: 2022

volume: 18

issue: 4

pages: 1016-1020

abstract: INTRODUCTION: Gigantomastia is a rare condition characterised by excessive breast growth. The pathophysiology of mammary enlargement varies depending on the type of gigantomastia: gestational, juvenile virginal, or idiopathic. The study aimed at examining the receptor status (oestrogen receptor α (ERα) and progesterone receptor (PR)) of breast tissue in adult women with juvenile or idiopathic gigantomastia. MATERIAL AND METHODS: The study involved 70 women who underwent breast reduction due to juvenile or idiopathic gigantomastia. Control breast specimens were obtained from 18 female cadavers. ERα and PR expressions were detected immunohistochemically in breast gland samples. RESULTS: Categorised and uncategorised ERα and PR expression did not differ between women with gigantomastia and control women. It was found that in both groups weak (0-30%) ERα and PR expression was the most common. Analysis of categorised data also did not reveal any significant correlations between ERα or PR and the women's age: for the whole group: $p = 0.795$ (ERα), $p = 0.207$ (PR), for women with gigantomastia: $p = 0.934$ (ERα), $p = 0.43$ (PR), and for control women: $p = 0.638$ (ERα), $p = 0.805$ (PR). CONCLUSIONS: Gigantomastia is not caused by increased expression of ERα and PR. Analysing abnormal sensitivity of these receptors to hormones may be crucial in establishing the increased risk of breast cancer in women with gigantomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9266871/

Record 698

pmcid: PMC9512736

pmid: 35043247

doi: 10.1007/s00266-021-02753-y

title: The Search for the Ideal Female Breast: A Nationally Representative United-States-Census Study.

authors

- Wallner C
- Dahlmann V
- Montemurro P
- Kümmel S
- Reinisch M
- Drysch M
- Schmidt SV
- Reinkemeier F
- Huber J
- Wagner JM
- Sogorski A
- Dadras M
- von Glinski M
- Lehnhardt M
- Behr B

journal: Aesthetic plastic surgery

journalAbbr: Aesthetic Plast Surg

year: 2022

pubdate: 2022 Aug

volume: 46

issue: 4

pages: 1567-1574

abstract: INTRODUCTION: Many studies have started to search for the perfect aesthetic breast in order to create a pars-pro-toto for reconstruction, but especially for aesthetic surgery. To date, no representative study with anatomically accurate models was performed. METHODS: In an online based United-States-census-representative survey with 1049 participants, questions regarding the preferred breast were asked utilizing lifelike morphed 3D-generated female models for the first time. Attributes such as breast pole ratio, areola size, breast direction and projection were asked. RESULTS: The results show that, contrary to what has been claimed in previous studies, an upper-pole-to-lower-pole ratio of 55:45 is preferred by both female and male participants. When it comes to breast size, on the other hand, there are clear gender-specific differences. While women opted for a cup size around B, the men preferred larger cup sizes. Moreover, the smallest depicted areola size of 30 mm was favored among all groups in the survey. DISCUSSION: Most publications used rather detrimental models for their surveys. We therefore opted for computer-generated 3D models and varied their naturalness. This enabled us to ensure a more aesthetic and accurate illustration and thus obtained more comparable and reliable results paired with the representation of the US-population. Taken together this study unveiled unexpected insights into the population favored breast attributes that might change operative planning in breast surgery. LEVEL OF EVIDENCE V: This journal requires that authors assign a level of evidence to each article. For a full description of these Evidence-Based Medicine ratings, please refer to the Table of Contents or the online Instructions to Authors http://www.springer.com/00266 .

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9512736/

Record 699

pmcid: PMC9552069

pmid: 36237260

doi: 10.21037/tcr-21-2841

title: Thematic trends and knowledge structure map of sentinel lymph node biopsy for breast cancer: a bibliometric analysis from 2010 to 2019.

authors

- Huo Y
- Fan T
- Chen S
- Liu Q
- Fang Y
- Yao F

journal: Translational cancer research

journalAbbr: Transl Cancer Res

year: 2022

pubdate: 2022 Sep

volume: 11

issue: 9

pages: 3092-3107

abstract: BACKGROUND: Breast cancer is the leading cancer-related deaths among women. Although great progress has been made in clinical surgical treatment, it is still urgently needed to look for a treatment model with smaller wounds, lower damage, and a better prognosis. Sentinel lymph node biopsy (SLNB) is a minimally invasive technique for breast cancer treatment, which can correctly assess the patients' condition, prognosis, and treatment response. METHODS: We performed a PubMed-based bibliometric analysis to investigate publication trends of SLNB in breast cancer and determined the annual distribution of annual publication numbers, countries, authors, languages, journals, and high-frequency major Medical Subject Headings (MeSH) terms. RESULTS: The results showed that the literature on SLNB in breast cancer has shown an upward trend, and stabilized with the most English literature in the past decade at least. The United States was the country with the most publications from 2010 to 2019. M Ahemd was the first-author who had published the most documents related to SLNB in breast cancer since 2010. The most high-frequency main MeSH words were breast neoplasms/pathology, breast neoplasms/surgery and SLNB. CONCLUSIONS: Through bicluster analysis, we divided the related articles of SLNB in breast cancer field from 2010 to 2019 into 4 clusters. Among them, indications for SLNB in breast cancer and detection of lymph node metastases and tracking methods for SLNB were considered to be current research hotspots, while assessment of axillary lymph nodes in neoadjuvant chemotherapy and application of SLNB was a potential hotspot.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9552069/

Record 700

pmcid: PMC9032001

pmid: 35455003

doi: 10.3390/life12040512

title: Thermal Boost to Breast Tumor Bed-New Technique Description, Treatment Application and Example Clinical Results.

authors

- Chicheł A
- Burchardt W
- Chyrek AJ
- Bielęda G
- Zwierzchowski G
- Stefaniak P
- Malicki J

journal: Life (Basel, Switzerland)

journalAbbr: Life (Basel)

year: 2022

pubdate: 2022 Mar 30

volume: 12

issue: 4

pages:

abstract: (1) Current breast-conserving therapy for breast cancer consists of a combination of many consecutive treatment modalities. The most crucial goal of postoperative treatment is to eradicate potentially relapse-forming residual cancerous cells within the tumor bed. To achieve this, the HDR brachytherapy boost standardly added to external beam radiotherapy was enhanced with an initial thermal boost. This study presents an original thermal boost technique developed in the clinic. (2) A detailed point-by-point description of thermal boost application is presented. Data on proper patient selection, microwave thermal boost planning, and interstitial hyperthermia treatment delivery are supported by relevant figures and schemes. (3) Out of 1134 breast cancer patients who were administered HDR brachytherapy boost in the tumor bed, 262 were also pre-heated interstitially without unexpected complications. The results are supported by two example cases of hyperthermia planning and delivery. (4) Additional breast cancer interstitial thermal boost preceding HDR brachytherapy boost as a part of combined treatment in a unique postoperative setting was feasible, well-tolerated, completed in a reasonable amount of time, and reproducible. A commercially available interstitial hyperthermia system fit and worked well with standard interstitial brachytherapy equipment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9032001/

Record 701

pmcid: PMC9118967

pmid: 34672482

doi: 10.5935/1518-0557.20210061

title: Tiger nut: Antidote for alcohol-induced testicular toxicity in male Sprague-Dawley rats.

authors

- Gbotolorun SC
- Salako AA
- Ogunlade B

journal: JBRA assisted reproduction

journalAbbr: JBRA Assist Reprod

year: 2022

pubdate: 2022 Apr 17

volume: 26

issue: 2

pages: 222-231

abstract: OBJECTIVE: Studies have implicated alcohol consumption as a factor leading to male infertility. Tiger nuts (Cyperus esculentus) on the other hand, have been shown to possess the potential to boost male reproductive indices. This study was carried out to investigate the effect of tiger nuts on alcohol-induced testicular toxicity in male Sprague-Dawley rats. METHODS: Thirty male Sprague-Dawley rats (160g averagely) were divided into six groups (A-F) (n=5). Group A (Control) received distilled water only; Group B (Tiger nut alone) received 1.8 g/kg body weight (BW) tiger nut; Groups C-F received 1 ml of 30% v/v alcohol three times weekly but groups C-E, also, received varied doses of tiger nut (0.6, 1.2 and 1.8 g/kg) (low, medium and high doses, respectively). All administrations were by oral gavage for 8 weeks. Serum was obtained and assayed for FSH, LH, and testosterone. Sperm was analyzed for semen parameters, and vitamins C and E contents. The testes were analyzed for antioxidants and histomorphology. RESULTS: There was a significant decrease in body and testicular weights, semen characteristics with altered contents of vitamins C and E, hormone profiles, and testicular morphology in alcohol-exposed rats when compared with the control animals. However, the administration of tiger nuts improved the testicular architecture, semen parameters, and antioxidant enzymes in a dose-dependent manner. CONCLUSIONS: Supplementation with tiger nuts following alcohol administration produces a reversal of the deleterious effect of alcohol on the testis in a dose-dependent manner.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9118967/

Record 702

pmcid: PMC8966063

pmid: 35371651

doi: 10.7759/cureus.22676

title: Transformation of Peripheral Sexual Precocity to Central Sexual Precocity Following Treatment of Granulosa Cell Tumor of the Ovary.

authors

- Gaikwad PM
- Goswami S
- Sengupta N
- Baidya A
- Das N

journal: Cureus

journalAbbr: Cureus

year: 2022

pubdate: 2022 Feb

volume: 14

issue: 2

pages: e22676

abstract: Juvenile granulosa cell tumor leading to isosexual peripheral precocious puberty is a well-known association. Here, we report a rare case of central precocious puberty secondary to granulosa cell tumor of the ovary. A five-year and five-month-old girl presented with a history of progressive enlargement of bilateral breasts and intermittent vaginal spotting, associated with growth acceleration. Elevated estradiol and suppressed serum follicle-stimulating hormone were found on investigation. Additionally, abdominal and pelvic ultrasonography was suggestive of a right ovarian mass, which proved to be a juvenile granulosa cell tumor on histopathology and immunohistochemistry, leading to a diagnosis of peripheral precocious puberty secondary to granulosa cell tumor of the ovary. One and a half years after resection of the tumor, secondary sexual characteristics progressed with regression of tumor markers, and no mass was noted on ultrasonography, leading to the

suspicion of central precocious puberty. Pubertal basal luteinizing hormone (LH) and elevated triptorelin-stimulated LH confirmed the diagnosis of central precocious puberty secondary to granulosa cell tumor of the ovary.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8966063/

Record 703

pmcid: PMC10226351

pmid: 35193333

doi: 10.5114/pedm.2021.109267

title: Ultrasonic evaluation of thelarche grading in 2-8-year-old obese girls suspected of precocious pubert.

authors

- Sadat Hosseini Z
- Shiasi Arani K
- Moraveji A
- Reza Talari H

journal: Pediatric endocrinology, diabetes, and metabolism

journalAbbr: Pediatr Endocrinol Diabetes Metab

year: 2022

pubdate: 2022

volume: 28

issue: 1

pages: 23-29

abstract: INTRODUCTION: The prevalence of obesity in children is increasing. In obese children, clinical examination alone is not sufficient to differentiate lipomastia from thelarche. The aim of this study was to investigate the frequency of true thelarche in obese girls suspected of precocious puberty. MATERIAL AND METHODS: In a cross-sectional study, 100 obese girls between the ages of 2 and 8 years were screened. Maturity stages were determined based on the Tanner scale in the questionnaire. Breast ultrasound was performed for all cases. Other uterine and ovarian laboratory tests, including blood tests to determine plasma levels of LH, FSH, oestradiol, and bone age, were also performed. RESULTS: In ultrasonography examination of 80 children (80%), one breast was pubertal and in 72 people both breasts were pubertal in which ultrasonography of 78 children showed puberty of right breast and 74 children showed puberty of left breast. Twenty children showed bilateral lipomastia. In clinical examination, breasts of 18.9% children seemed lipoid, 35.8% children were seriously suspicious, and 45.3% children seemed pubescent. In pelvic ultrasound, the relationship between ovarian volume and breast ultrasound grade was significant, but uterine volume was not significantly related to breast ultrasound grade. Also, uterine volume with age at ultrasonography, bone age, birth height, left ovarian volume, right ovarian volume, right breast bud diameter, and left breast bud diameter were correlated. CONCLUSIONS: Ultrasound can help obese or overweight children to differentiate between true thelarche and lipomastia. The study also found that most suspected children had true thelarche.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10226351/

Record 704

pmcid: PMC9309312

pmid: 35898923

doi: 10.2147/PPA.S366522

title: Understanding the Treatment Preferences of People Living with Schizophrenia in Australia; A Patient Value Mapping Study.

authors

- Fifer S
- Keen B
- Newton R
- Puig A
- McGeachie M

journal: Patient preference and adherence

journalAbbr: Patient Prefer Adherence

year: 2022

pubdate: 2022

volume: 16

issue:

pages: 1687-1701

abstract: PURPOSE: To examine the treatment and long-term outcome preferences for people living with schizophrenia. PATIENTS AND METHODS: Sixty-six Australian adults, living with schizophrenia completed a novel online survey with six sections: Demographic characteristics; Disease history; Quality-of-life; Patient support programmes; Discrete Choice Experiment, and Best-Worst Scaling exercise. RESULTS: Participants indicated that they preferred to be involved in treatment decision with their doctor. A minority of participants reported having been previously involved in a patient support programme (28.8%) and only one in six participants had a National Disability Insurance Scheme (NDIS) package (16.7%) with over a third of participants indicating that they were ineligible (37.9%). Participants' average quality-of-life score was 60%. CONCLUSION: Recent hospitalisation influenced the relative importance of treatment attributes, with effectiveness on hearing voices being the most important treatment attribute. The most important long-term goals were having a stable place to live, being independent, and physical health. People with schizophrenia care about their long-term functional recovery outcomes, rating symptom control and independence as their highest priority. They want to be part of the treatment conversation with their doctors. Therefore, psychiatrists are encouraged to use shared decision-making to establish the treatment course that best aligns with individuals' long-term goals.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9309312/

Record 705

pmcid: PMC11195659

pmid: 38919779

doi: 10.5737/23688076324512

title: Unmet supportive care needs in prostate cancer survivors with advanced disease: A mixed-methods exploration.

authors

- Calvo-Schimmel A
- Newman SD
- Sterba KR
- Mueller M
- Miaskowski C
- Qanungo S

journal: Canadian oncology nursing journal = Revue canadienne de nursing oncologique

journalAbbr: Can Oncol Nurs J

year: 2022

pubdate: 2022 Fall

volume: 32

issue: 4

pages: 512-525

abstract: PURPOSE: Men with advanced prostate cancer experience a wide range of side effects from the cancer and its therapies, which have a negative effect on their quality of life (QOL). Few studies have evaluated supportive care needs in these individuals. The purpose of this study was to conduct a holistic supportive care needs assessment among these survivors guided by the Supportive Care Framework for Cancer Care. METHODS: Using a convergent parallel mixed-methods approach, prostate cancer survivors with advanced disease (n = 188) completed a cross-sectional survey. A subset of these survivors (n = 20) participated in an interview to further explore their experience of unmet needs. RESULTS: Survivors reported unmet supportive care needs in every domain of the framework. Up to 95.2% of the survivors had at least one unmet need, with a mean of 14.9 (range: 0-42). Several areas of convergence among the quantitative and qualitative data (fatigue, sexual dysfunction, practical, and emotional/psychological domains), as well as divergence (informational and spiritual domains, depression, urinary dysfunction) were found through the integration process. CONCLUSIONS: This study confirms that prostate cancer survivors with advanced disease experience high rates of unmet supportive care needs. The findings also highlight the diversity of those unmet needs. These results may assist with future development of patient-centered supportive care interventions that better meet the specific needs of this vulnerable group of cancer survivors.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11195659/

Record 706

pmcid: PMC9086347

pmid: 35534426

doi: 10.6118/jmm.21028

title: Vaginal Vitamin E for Treatment of Genitourinary Syndrome of Menopause: A Systematic Review of Randomized Controlled Trials.

authors

- Porterfield L
- Wur N
- Delgado ZS
- Syed F
- Song A
- Weller SC

journal: Journal of menopausal medicine

journalAbbr: J Menopausal Med

year: 2022

pubdate: 2022 Apr

volume: 28

issue: 1

pages: 9-16

abstract: Genitourinary syndrome of menopause significantly affects the quality of life in postmenopausal women with few evidence-based alternatives to vaginal estrogen for women with contraindications. This systematic review evaluates the evidence for vaginal vitamin E efficacy in reducing patient-reported

genitourinary symptoms in healthy postmenopausal women compared to placebo or vaginal control therapy. This systematic review evaluated randomized controlled trials before October 2020 that assessed the efficacy of vitamin E vaginal suppositories in reducing genitourinary symptoms in postmenopausal women compared with a control group of healthy postmenopausal women. Outcomes included patient-perceived genitourinary symptoms. Of the 31 studies, four met the inclusion criteria. One 8-week trial (n = 42) found a significant reduction in vaginal symptoms in the 1 mg vitamin E group than the placebo group (difference in means, 5.3; 95% confidence interval [CI], 4.4 to 6.2). Another 8-week trial (n = 40) found 5 mg vaginal hyaluronic acid superior to 1 mg vitamin E (difference in means -0.50, 95% CI, -0.95 to -0.05). Two 12-week trials (n = 52 in each) found no difference between 0.5 g vaginal estrogen and 100 IU vaginal vitamin E in healthy postmenopausal women (difference in means: -0.19, 95% CI, -4.4 to 4.0, and -3.47, 95% CI, -13.8 to 6.8). Evidence from small, limited studies suggests that vaginal vitamin E may be effective in alleviating symptoms of genitourinary syndrome of menopause; however, additional high-quality studies are needed to determine efficacy, ideal dosing, and long-term safety.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9086347/

Record 707

pmcid: PMC8733184

pmid: 35024488

doi: 10.1016/j.heliyon.2021.e08668

title: Wandering through southwestern Nigeria: An inventory of Yoruba useful angiosperm plants.

authors

- Ajao AA
- Mukaila YO
- Sabiu S

journal: Heliyon

journalAbbr: Heliyon

year: 2022

pubdate: 2022 Jan

volume: 8

issue: 1

pages: e08668

abstract: This paper is a compilation of all known uses of angiosperm plants by the Yoruba people of southwestern Nigeria. Information was gathered from the past experiences of authors and surveys of books, journal articles, dissertations (published and unpublished) and theses using online databases. The review presents 493 angiosperm species (65 monocots and 428 dicots) belonging to 99 families, of which Fabaceae contributed the highest number of useful plants (72 spp.), followed by Euphorbiaceae (31 spp.), Malvaceae (30 spp.), and Asteraceae (25 spp.). Generally, of the identified plants, 418 species are for medicinal purposes, 85 species are utilized as food and beverages, 65 species for other uses including games, food packaging, and arts and crafts while 22 species are used for magical purposes such as success charm, enhancing disappearance, protection from witches and escaping from the repercussion of an act. This study provides baseline ethnobotanical data for future quantitative analyses of useful plants in the region, as indigenous plant knowledge has not been properly explored and documented among the Yoruba people.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8733184/

Record 708

pmcid: PMC9549314

pmid: 36247337

doi: 10.1002/agm2.12223

title: We are all aging, and here's why.

authors

- Shinde A
- Deore G
- Navsariwala KP
- Tabassum H
- Wani M

journal: Aging medicine (Milton (N.S.W))

journalAbbr: Aging Med (Milton)

year: 2022

pubdate: 2022 Sep

volume: 5

issue: 3

pages: 211-231

abstract: Here, through this review, we aim to serve this purpose by first discussing the statistics and aging demographics, including the life expectancy of the world and India, along with the gender life expectancy gap observed throughout the world, followed by explaining the hallmarks and integral causes of aging, along with the role played by senescent cells in controlling inflammation and the effect of senescence associated secretory phenotype on longevity. A few of the molecular pathways which are crucial in modulating the process of aging, such as the nutrient-sensing mTOR pathway, insulin signaling, Nrf2, FOXO, PI3-Akt, Sirtuins, and AMPK, and their effects are also covered in paramount detail. A diverse number of ingenious research methodologies are used in the modern era of longevity exploration. We have attempted to cover these methods under the umbrella of three broad categories: in vitro, in vivo, and in silico techniques. The drugs developed to attenuate the aging process, such as rapamycin, metformin, resveratrol, etc. and their interactions with the above-mentioned molecular pathways along with their toxicity have also been reviewed in detail.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9549314/

Record 709

pmcid: PMC8299094

pmid: 34322243

doi: 10.1002/ccr3.4444

title: A case of severe side effect due to muscle oil injections in combination with chemotherapy in a former bodybuilder.

authors

- Gitz-Johansen G
- Als AB
- Pallesen KAU

journal: Clinical case reports

journalAbbr: Clin Case Rep

year: 2021

pubdate: 2021 Jul

volume: 9

issue: 7

pages: e04444

abstract: This case report presents an acute painful skin reaction several years after injection of site-enhancing oils (SEO) induced by chemotherapy. This case exemplifies the long-term dangerous side effects of the use of SEO, however treatable with prednisolone.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8299094/

Record 710

pmcid: PMC8477099

pmid: 34487069

doi: 10.23750/abm.v92i4.11572

title: A rare case of Ovarian Juvenile Granulosa Cell Tumor in an Infant with Isosexual Pseudo Puberty and a Revision of Literature.

authors

- De Sanctis V
- Ahmed S
- Soliman A
- Alyafei F
- Alaaraj N
- Al Maadheed M
- Clelland C

journal: Acta bio-medica : Atenei Parmensis

journalAbbr: Acta Biomed

year: 2021

pubdate: 2021 Sep 2

volume: 92

issue: 4

pages: e2021319

abstract: Juvenile ovarian granulosa cell tumors (JGCTs) are described infrequently in pediatrics, and their finding in infants is exceptional. We highlight the presenting symptoms, radiologic images, operative management, and histopathologic findings of a 9-month-old female with isosexual pseudopuberty. An updated revision of literature in infants below the age of 12 months is also reported.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8477099/

Record 711

pmcid: PMC10227485

pmid: 33599439

doi: 10.5114/pedm.2020.101803

title: A rare case of primary amenorrhoea and breast development in a 46,XY 15-year-old girl.

authors

- Morawiecka-Pietrzak M
- Dąbrowska E

- Gliwińska A
- Góra A
- Geisler G
- Gawlik A
- Kudela G
- Ziora K

journal: Pediatric endocrinology, diabetes, and metabolism

journalAbbr: Pediatr Endocrinol Diabetes Metab

year: 2021

pubdate: 2021

volume: 27

issue: 1

pages: 62-67

abstract: A disorder of sex development (DSD) is defined as a congenital condition in which development of chromosomal, gonadal, or anatomical sex is atypical. Swyer syndrome is an example of 46,XY DSD with a female phenotype. It usually becomes apparent in adolescence with delayed puberty and amenorrhoea. Spontaneous breast development is very rare. A 15-year-old girl was presented due to primary amenorrhoea with breast development compatible with Tanner stage V. Hormonal tests revealed hypergonadotropic hypogonadism with low level of oestradiol. Pelvic ultrasound and magnetic resonance imaging revealed a small uterus, and no ovaries were found. In the right lower abdomen, a structure of unknown origin was visible. The chromosome analysis revealed a 46,XY karyotype. The patient was qualified for a laparoscopic bilateral gonadectomy. Postoperative histopathological examination revealed gonadoblastoma. We underline the need to consider DSD 46,XY in the presence of primary amenorrhoea, even when pubertal development is present. Germ cell tumors have a tendency to grow and metastasize rapidly. Delayed diagnosis may increase the risk of malignant transformation and cause a poor diagnosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10227485/

Record 712

pmcid: PMC7882310

pmid: 33634000

doi: 10.21037/gs.2020.03.06

title: Advising patients about breast implant associated anaplastic large cell lymphoma.

authors

- Parham CS
- Hanson SE
- Butler CE
- Calobrace MB
- Hollrah R
- Macgregor T
- Clemens MW

journal: Gland surgery

journalAbbr: Gland Surg

year: 2021

pubdate: 2021 Jan

volume: 10

issue: 1

pages: 417-429

abstract: The United States Food and Drug Administration (FDA) recently recognized the necessity for improved patient education and public awareness of breast implant-associated anaplastic large cell lymphoma (BIA-ALCL). BIA-ALCL is an uncommon T-cell lymphoma that can arise years after placement of a textured breast implant or tissue expander. Patients and providers would greatly benefit from a broader awareness of BIA-ALCL and how it presents with both common and rare symptoms. Disease awareness can be improved with specific steps moving forward. This includes improved counseling about annual recommendations from the FDA, standard of care guidelines, current estimates of disease prevalence, and manufacturer-specific risk for developing the malignancy. Informed consent for implant-based breast surgery should include a discussion of BIA-ALCL, symptoms, inherent risks and alternatives. Consideration should be given for reasonable efforts to retroactively contact patients with textured implants to inform them of recent regulatory actions as well as the risk of developing the disease and the signs and symptoms to watch for. Where risk can be reduced, all steps should be taken to ensure patient safety. In keeping with our commitment to patient safety, the strategies moving forward for implant-based breast surgery should be marked by patient awareness, physician vigilance, and defined by evidence-based diagnosis and treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7882310/

Record 713

pmcid: PMC8514307

pmid: 34733731

doi: 10.21037/gs-20-892

title: Aesthetic breast surgery: putting in context-a narrative review.

authors

- Roy PG
- Yan Z
- Nigam S
- Maheshwari K

journal: Gland surgery

journalAbbr: Gland Surg

year: 2021

pubdate: 2021 Sep

volume: 10

issue: 9

pages: 2832-2846

abstract: OBJECTIVE: This review article summarises the latest evidence for commonly undertaken procedures in aesthetic breast surgery with a focus on key principles of breast augmentation, reduction, and mastopexy. The paper also outlines various approaches and controversies as well as complications such as breast implant associated anaplastic large cell lymphoma (BIA-ALCL) and breast implant illness (BII) which are increasingly being recognised and becoming a challenge to manage. BACKGROUND: Changing trends of aesthetic breast surgery over the decades has warranted a continuous evolution of this field. The ability to deliver safe and appropriate care is dependent upon sound reconstructive principles and proper training. The lack of uniformity in either is a cause of concern. The impact of social media and changing perception of body image can also no longer be overlooked in the field of aesthetics and reconstruction. METHODS: Review of literature including recent journals, textbook chapters, online databases like PubMed, and current government and surgical society guidelines. CONCLUSIONS: Breast reconstruction is based on sound surgical principles and it is imperative to

follow these for the practice of this speciality. There are two important issues that revolve around this aspect of surgery. First relates to the urgent need to invest time and effort in improving regulations and outcomes in the cosmetic surgery industry. Second, it is crucial to promote and prioritize the development and training in this field as the principles of aesthetic breast surgery underpins oncoplastic breast surgery for management of cancer.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8514307/

Record 714

pmcid: PMC8279075

pmid: 34278180

doi: 10.1210/jendso/bvab068

title: An Approach to Nonsuppressed Testosterone in Transgender Women Receiving Gender-Affirming Feminizing Hormonal Therapy.

authors

- Maheshwari A
- Nippoldt T
- Davidge-Pitts C

journal: Journal of the Endocrine Society

journalAbbr: J Endocr Soc

year: 2021

pubdate: 2021 Sep 1

volume: 5

issue: 9

pages: bvab068

abstract: Nonsuppressed levels of testosterone are seen in up to a quarter of transgender women on gender-affirming feminizing hormonal treatment. Multiple factors contribute to this situation, including patient, medication, laboratory, and organ-specific concerns. We propose a stepwise approach to determine the etiology of nonsuppressed levels of testosterone in transgender women. This may lead to an appropriate feminizing hormonal therapy regimen and diagnosis of manageable medical conditions.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8279075/

Record 715

pmcid: PMC7915286

pmid: 33572240

doi: 10.3390/antibiotics10020172

title: An Evaluation of Antibiotic Prescribing Practices in a Rural Refugee Settlement District in Uganda.

authors

- Bonniface M
- Nambatya W
- Rajab K

journal: Antibiotics (Basel, Switzerland)

journalAbbr: Antibiotics (Basel)

year: 2021

pubdate: 2021 Feb 9

volume: 10

issue: 2

pages:

abstract: Ensuring access to effective antibiotics and rational prescribing of antibiotics are critical in reducing antibiotic resistance. In this study, we evaluated antibiotic prescribing practices in a rural district in Uganda. It was a cross-sectional study that involved a retrospective review of 500 outpatient prescriptions from five health facilities. The prescriptions were systematically sampled. World Health Organization core medicine use prescribing and facility indicators were used. Percentage of encounters with one or more antibiotics prescribed was 23% (10,402/45,160). The mean number of antibiotics per prescription was 1.3 (669/500). About 27% (133/500) of the diagnoses and 42% (155/367) of the prescriptions were noncompliant with the national treatment guidelines. Prescribing antibiotics for nonbacterial infections such as malaria 32% (50/156) and noninfectious conditions such as dysmenorrhea and lumbago 15% (23/156) and nonspecific diagnosis such as respiratory tract infection 40% (59/133) were considered noncompliant with the guidelines. On average, 68% (51/75) of the antibiotics were available on the day of the visit. Inappropriate prescribing practices included excessive use of antibiotics and failure to diagnose and prescribe in compliance with treatment guidelines. There is a need to strengthen antibiotic use in the health facilities through setting up stewardship programs and interventions to promote adherence to national treatment guidelines.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7915286/

Record 716

pmcid: PMC8306633

pmid: 34356984

doi: 10.3390/medicina57070703

title: Anabolic-Androgenic Steroid Abuse among Gym Users, Eastern Province, Saudi Arabia.

authors

- Albaker W
- Alkhars A
- Elamin Y
- Jatoi N
- Boumarah D
- Al-Hariri M

journal: Medicina (Kaunas, Lithuania)

journalAbbr: Medicina (Kaunas)

year: 2021

pubdate: 2021 Jul 10

volume: 57

issue: 7

pages:

abstract: Background and Objectives : The main aim of the present study was to assess the use of androgenic-anabolic steroids (AAS) and to investigate its potentially unfavorable effects among gym members attending gym fitness facilities in Eastern Province, Saudi Arabia. Materials and Methods : A cross-sectional questionnaire-based study was carried out during the summer of 2017. Male gym users in the Eastern Province region of Saudi Arabia were the respondents. Information on socio-demographics, use of AAS, knowledge, and awareness about its side effects were collected using a self-administered questionnaire. Results: The prevalence of AAS consumption among trainees in Eastern Province was 21.3%. The percentage was highest

among those 26-30 years of age (31.9%), followed by the 21-25 (27.4%) (p = 0.003) age group. Participants in the study were not aware of the potential adverse effects of AAS use. Adverse effects experienced by 77% of AAS users include psychiatric problems (47%), acne (32.7%), hair loss (14.2%), and sexual dysfunction (10.7%). Moreover, it appears that trainers and friends are major sources (75.20%) for obtaining AAS. Conclusion: AAS abuse is a real problem among gym members, along with a lack of knowledge regarding its adverse effects. Health education and awareness programs are needed not only for trainees, but also for trainers and gym owners as they are reportedly some of the primary sources of AAS.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8306633/

Record 717

pmcid: PMC8531735

pmid: 34760808

doi: 10.4103/abr.abr_298_20

title: Anaplastic Large Cell Lymphoma, Giant Cell-Rich, Involving a Nonimplant Breast: A Case Report and Review of the Literature.

authors

- Al-Zaidi RS
- Al-Noor NI

journal: Advanced biomedical research

journalAbbr: Adv Biomed Res

year: 2021

pubdate: 2021

volume: 10

issue:

pages: 26

abstract: Primary breast lymphomas are uncommon tumors and account for <1% of all malignant neoplasms of the breast. They are almost always of non-Hodgkin type, with B-cell lymphomas being the most common subtype. Anaplastic large cell lymphoma (ALCL) is a rare T-cell lymphoma that can involve the breast. Most of the articles in the literature describe ALCL in association with breast implants. We present a 48-year-old woman with a left breast enlargement and no history of an implant. Microscopic sections showed a high-grade CD30-positive lymphoid neoplasm with frequent giant cells, which turned out to be a primary ALCL of the breast, giant cell-rich pattern. To our knowledge, no cases of primary ALCL, giant cell-rich variant, have been reported in the breast in the absence of an implant making our case unique.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8531735/

Record 718

pmcid: PMC8033579

pmid: 33827127

doi: 10.2344/anpr-68-01-15

title: Anesthetic Management of the Pregnant Patient: Part 1.

authors

- Shin J

journal: Anesthesia progress

journalAbbr: Anesth Prog

year: 2021

pubdate: 2021 Mar 1

volume: 68

issue: 1

pages: 52-62

abstract: As delays in the age for a mother's first pregnancy continue to trend upward globally, particularly in developed countries, many pregnant patients are increasingly educated on the importance of obtaining dental care throughout their pregnancies. Guidelines set forth by the American Dental Association and the American College of Obstetrics and Gynecologists highlight the importance of dental treatment for optimizing maternal-fetal health across all trimesters, especially for emergent dental issues. The pregnant dental patient undergoes significant physiologic remodeling unique to each trimester, which may complicate treatment. Providing safe anxiety and pain control for dentistry can be further complicated if sedation or general anesthesia is required for the parturient. This is even more true when superimposed with increasingly prevalent underlying comorbidities like hypertension and diabetes. As dental providers, there is a clear need for continuing education on the many challenges associated with caring for pregnant patients due to this being an often overlooked subject in undergraduate and postgraduate dental education. Part 1 of this review will present the maternal and fetal physiologic considerations and the impact on patient management from an anesthetic perspective. Additional discussion focusing on common sedative and anesthetic agents used during dental procedures and their considerations will follow in Part 2.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8033579/

Record 719

pmcid: PMC8591998

pmid: 34790067

doi: 10.5863/1551-6776-26.8.783

title: Antiretroviral Therapy in Children and Adolescents: A Look Into Modern Single Tablet Regimens.

authors

- Lee C
- Sapasap J
- LaRochelle J
- Smith RO
- Badowski ME

journal: The journal of pediatric pharmacology and therapeutics : JPPT : the official journal of PPAG

journalAbbr: J Pediatr Pharmacol Ther

year: 2021

pubdate: 2021

volume: 26

issue: 8

pages: 783-794

abstract: Single tablet regimens (STRs) have simplified antiretroviral therapy (ART) over the years in the adult human immunodeficiency virus (HIV) population. However, there is still a prevalent need to simplify regimens in children and adolescents living with HIV. Finding the optimal regimen requires a multi-factorial approach due to their complex pharmacokinetic profiles throughout childhood and the challenges and limitations of medication non-adherence in the pediatric population. These challenges include pill size, available formulations, palatability, and caregiver health literacy, which can all affect the proper administration of medications. The complexity of this population implies the importance of customizing everyone's antiretroviral regimen so that the patient and

family can successfully adhere to the therapy. The current recommendations for ART in the adult and pediatric populations are similar, yet the use of STRs are limited. The goal of this review was to assess current data on available STRs and determine their utility as ART in the pediatric population.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8591998/

Record 720

pmcid: PMC7995469

pmid: 33796446

doi: 10.1016/j.crwh.2021.e00308

title: Aripiprazole and lactation failure: The importance of shared decision making. A case report.

authors

- Komaroff A

journal: Case reports in women's health

journalAbbr: Case Rep Womens Health

year: 2021

pubdate: 2021 Apr

volume: 30

issue:

pages: e00308

abstract: Bipolar disorder is a chronic and severe psychiatric illness affecting many patients during their childbearing years. Bipolar disorder is often managed with aripiprazole, a generally well tolerated second-generation antipsychotic medication. Published data regarding its safety profile are reassuring and aripiprazole is prescribed during pregnancy, postpartum and during lactation. Pregnancy and the postpartum period represent times of increased vulnerability for patients with bipolar disorder, especially those who are untreated, highlighting the need for medical management. However, aripiprazole may interfere with human milk production. Currently, there is limited evidence to understand lactation failure in patients who take aripiprazole. This case reinforces the need for shared decision making regarding the potential impact on lactation when aripiprazole is being considered for the pregnant patient with bipolar disorder.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7995469/

Record 721

pmcid: PMC8176602

pmid: 34082831

doi: 10.1186/s13256-021-02897-0

title: Atraumatic thoracic spinal fracture mimicking herpes zoster neuralgia: a case report.

authors

- Cao L
- Xiao X
- Du S

journal: Journal of medical case reports

journalAbbr: J Med Case Rep

year: 2021

pubdate: 2021 Jun 4

volume: 15

issue: 1

pages: 301

abstract: BACKGROUND: Intercostal neuralgia is most common in patients with herpes zoster, but it might be the initial symptom of serious diseases, such as atraumatic spinal fracture, which may lead to serious consequences if not diagnosed and treated early. Severe intercostal neuralgia is rarely reported as the first symptom of ankylosing spondylitis with atraumatic vertebral fractures. CASE PRESENTATION: A 70-year-old Chinese Han man previously diagnosed with ankylosing spondylitis presented to the hospital with intense intercostal pain without trauma. The patient was initially suspected of having herpes zoster neuralgia; however, he subsequently experienced numbness and weakness of both lower limbs as well as constipation. Thoracic vertebral fracture and compression of the spinal cord were detected with magnetic resonance imaging, and he underwent emergency posterior thoracic spinal canal decompression, and intercostal neuralgia was relieved after surgery. Spinal tuberculosis and tumors were later excluded by pathological examination and follow-up results. A 6-month postoperative follow-up showed that the weakness and numbness of the left lower limb had significantly improved, and his urinary function had recovered. CONCLUSIONS: Patients with ankylosing spondylitis could develop atraumatic spinal fractures. Severe intercostal neuralgia is an early indicator of spinal fractures, and spinal magnetic resonance imaging is crucial for the diagnosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8176602/

Record 722

pmcid: PMC8351151

pmid: 34365967

doi: 10.1186/s13256-021-02996-y

title: Becker's nevus syndrome: a case report.

authors

- Chikani UN
- Ohuche IN
- Bisi-Onyemaechi AI

journal: Journal of medical case reports

journalAbbr: J Med Case Rep

year: 2021

pubdate: 2021 Aug 9

volume: 15

issue: 1

pages: 395

abstract: BACKGROUND: Becker's nevus syndrome is a syndrome characterized by the presence of a Becker's nevus with ipsilateral breast hypoplasia or hypoplastic defects of the muscle, skin, or skeleton. The nevus usually consists of a circumscribed, unilateral, irregularly shaped hyperpigmented macule, commonly occurring around the anterior upper trunk, with/without hypertrichosis and/or acneiform lesions. This rare syndrome has not been reported in our locality to the best of our knowledge. CASE PRESENTATION: We report the case of a 15-year-old Igbo female patient who presented to our pediatric endocrinology clinic, University of Nigeria Teaching Hospital, Enugu, with complaints of asymmetry of the breasts and hyperpigmented macules on the side. Based on her symptoms, diagnosis of Becker's nevus syndrome was made. The diagnosis of Becker's nevus syndrome is mostly clinical, based on the presence of a Becker's nevus with ipsilateral breast hypoplasia or hypoplastic defects of the muscle, skin, or skeleton. In our patient, there was a Becker's nevus with ipsilateral breast hypoplasia. This syndrome, belonging to the class of epidermal nevus syndromes, is very rare, and is usually benign. She was placed on spironolactone tablets 50 mg daily, which have been associated with an

improvement in the size of the hypoplastic breast, and her fears were allayed. CONCLUSION: This syndrome has not been reported in our locality to the best of our knowledge and, therefore, has a propensity for misdiagnosis by clinicians because of its rarity. We therefore report this to create awareness among clinicians regarding this condition that is associated with much psychosocial trauma among patients, and that can be easily managed with oral spironolactone.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8351151/

Record 723

pmcid: PMC8238392

pmid: 34160454

doi: 10.1097/MD.0000000000026469

title: Bilateral breast myxedema caused by Graves' disease and responsive to multipoint subcutaneous injection of long-acting glucocorticoid: Case report.

authors

- Du T
- Jiang W
- Luo H
- Chen F
- Yuan G
- Zhang M
- Liu Z

journal: Medicine

journalAbbr: Medicine (Baltimore)

year: 2021

pubdate: 2021 Jun 25

volume: 100

issue: 25

pages: e26469

abstract: RATIONALE: With the absence of ophthalmopathy, thyroid dermopathy especially lesions at atypical locations is a very rare presentation. We herein report an original case of bilateral breast myxedema caused by Grave's disease. PATIENT CONCERNS: A 21-year-old unmarried woman presented with a 4-month history of Grave's disease and a 1-month history of progressive bilateral breast enlargement. She had symmetrical bilateral breast enlargement with redness and nonpitting thickening of the skin, diffusely enlarged thyroid glands, and no exophthalmos. DIAGNOSIS: Ultrasonography, magnetic resonance imaging scan, and skin biopsy confirmed the diagnosis of bilateral breast myxedema. INTERVENTIONS: The patient was treated with multipoint subcutaneous injections of triamcinolone acetonide in each breast every month. OUTCOMES: The bilateral breast returned approximately to its normal size after therapy for 6 months. CONCLUSIONS: Our case illustrates that multipoint subcutaneous injection of glucocorticoids is beneficial for bilateral breast myxedema.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8238392/

Record 724

pmcid: PMC8346061

pmid: 34360498

doi: 10.3390/ijerph18158206

title: Biopsychosocial Factors during the Perinatal Period: Risks, Preventative Factors, and Implications for

Healthcare Professionals.

authors

- Blount AJ
- Adams CR
- Anderson-Berry AL
- Hanson C
- Schneider K
- Pendyala G

journal: International journal of environmental research and public health

journalAbbr: Int J Environ Res Public Health

year: 2021

pubdate: 2021 Aug 3

volume: 18

issue: 15

pages:

abstract: Women face risks to their wellbeing during the perinatal period of pregnancy. However, there is a dearth of information on perinatal risk factors within the biopsychosocial paradigm. Emphasis is often placed on biological components associated with pregnancy and women's health. However, psychological and social determinants of health are integral during the perinatal period, and mental wellness is often a determinant for positive maternal and neonatal health outcomes. This article reviews risk factors of perinatal wellness (e.g., physical and nutritional concerns, trauma, discrimination, adverse childhood events) and highlights protective factors for women in their perinatal period. Healthcare professionals can support perinatal health by focusing on culturally and contextually appropriate research and prevention, providing equal access to sexual and reproductive healthcare information and services, providing quality education and training for helping professionals, and supporting policies for positive sexual and reproductive women's healthcare.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8346061/

Record 725

pmcid: PMC8408432

pmid: 34484543

doi: 10.1016/j.radcr.2021.08.006

title: Bloody nipple discharge caused by an intraductal papilloma of the breast in an adolescent girl: A case report.

authors

- Lee JY

journal: Radiology case reports

journalAbbr: Radiol Case Rep

year: 2021

pubdate: 2021 Nov

volume: 16

issue: 11

pages: 3346-3349

abstract: Bloody nipple discharge in pediatric patients is rare and mostly associated with benign

conditions.Despite the generally benign nature, a thorough investigation of the cause and treatment is required if a palpable lesion is present. Here, the author reports a case of bloody nipple discharge in an adolescent girl with no significant medical history. Breast ultrasound demonstrated a solid, oval-shaped, circumscribed mass in the left subareolar region that was categorized as category 4a according to the breast imaging reporting and data system (BI-RADS). An excisional biopsy and histological examination confirmed a diagnosis of intraductal papilloma. While intraductal papilloma is rare in the pediatric population, ultrasound evaluation and knowledge of characteristic findings are useful for noninvasive diagnostics and image-guided treatment planning.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8408432/

Record 726

pmcid: PMC7882356

pmid: 33634001

doi: 10.21037/gs-20-231

title: Breast implant illness: a topic in review.

authors

- Kaplan J
- Rohrich R

journal: Gland surgery

journalAbbr: Gland Surg

year: 2021

pubdate: 2021 Jan

volume: 10

issue: 1

pages: 430-443

abstract: Recently, the term breast implant illness has become popularized in social media to describe a constellation of symptoms which have been attributed to a patient's breast implants. These symptoms include fatigue, chest pain, hair loss, headaches, chills, photosensitivity, rash, and chronic pain amongst others. While physicians aim to treat these physical symptoms, currently the evidence supports the safety of silicone breast implants. This article entitled "Breast implant illness: a topic in review" presents an up-to-date review focusing on the safety of silicone breast implants. Patients retain the right to decide to keep or remove their breast implants and for those who choose to pursue explantation, they should be advised to seek the care of a board-certified plastic surgeon. As a scientific community is our duty to continue to conduct well-designed scientific studies to gain more insight into the safety of breast implants as it related to cancer detection, autoimmune disease, and other health concerns to improve patient safety, awareness, and education. This review article aims to delineate both the content and timing of all research and evidence as it pertains to the newly coined phrase "breast implant illness". The authors of this study support that currently there have not been any concrete or evidence-based studies which support the formation of a new syndrome "silicone implant illness".

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7882356/

Record 727

pmcid: PMC8426943

pmid: 34405902

doi: 10.1002/jcu.23056

title: Breast implant seroma: A SARS-CoV-2 mRNA vaccine side effect.

authors

- Kayser F

- Fourneau H
- Mazy OC
- Mazy S

journal: Journal of clinical ultrasound : JCU

journalAbbr: J Clin Ultrasound

year: 2021

pubdate: 2021 Nov

volume: 49

issue: 9

pages: 984-986

abstract: Fluid collection surrounding breast implants, called seroma, is a frequent issue that has diverse etiologies. We describe a 48-year-old woman with an acute onset of bilateral breast swelling occurring 5 years after aesthetic surgery and 10 days after a second SARS-CoV-2 mRNA vaccine dose. At breast ultrasound, we found an abundant bilateral peri-implant seroma. Our patient noticed symptom improvement 48 h after nonsteroidal anti-inflammatory drug treatment, complete disappearance of symptoms after 10 days with seroma disappearance documented by ultrasound. We hypothesized a breast implant seroma as post-SARS-CoV-2 mRNA vaccine side effect, as different other diagnosis had been excluded.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8426943/

Record 728

pmcid: PMC8491802

pmid: 34659917

doi: 10.7759/cureus.18478

title: Breastfeeding Insufficiencies: Common and Preventable Harm to Neonates.

authors

- Wilde VK

journal: Cureus

journalAbbr: Cureus

year: 2021

pubdate: 2021 Oct

volume: 13

issue: 10

pages: e18478

abstract: Insufficient milk intake in breastfed neonates is common, frequently missed, and causes preventable hospitalizations for jaundice/hyperbilirubinemia, hypernatremia/dehydration, and hypoglycemia - accounting for most U.S. neonatal readmissions. These and other consequences of neonatal starvation and deprivation may substantially contribute to fully preventable morbidity and mortality in previously healthy neonates worldwide. Previous advanced civilizations recognized this problem of breastfeeding insufficiencies and had an infrastructure to solve it: Wetnursing, shared nursing, and prelacteal feeding traditions used to be well-organized and widespread. Modern societies accidentally destroyed that infrastructure. Then, modern reformers missing a few generations of direct knowledge transmission about safe breastfeeding invented a new, historically anomalous conception of breastfeeding defined in terms of exclusivity. As that new intervention has become increasingly widespread, so too have researchers widely reported associated possible harms of the longer neonatal starvation/deprivation and later infant under-nutrition periods that it creates when breastfeeding is insufficient. Early insufficient nutrition/hydration has possible long-term effects including neurodevelopmental

consequences such as attention deficit hyperactivity disorder, autism, cerebral palsy, cognitive and developmental delay, epilepsy, hearing impairment, kernicterus, language disorder, mood disorders, lower IQ, and specific learning disorder. Current early infant feeding guidelines conflict with the available evidence. Recent reform efforts have tended to focus on using more technology and measurement to harm fewer neonates instead of proposing the indicated paradigm shift in early infant feeding to prevent more harm. The scientific evidence is already sufficient to mandate application of the precautionary principle to feed neonates early, adequate, and often milk before mothers' milk comes in and whenever signs of hunger persist, mitigating possible risks including death or disability. In most contexts, the formula is the best supplementary milk for infants at risk from breastfeeding insufficiencies. National-level reviews of scientific evidence, health policy, and research methods and ethics are needed to initiate the early infant feeding paradigm shift that the data already support. Policy experiments and related legislative initiatives might also contribute to the shift, as insurers might decline or be required by law to decline reimbursing hospitals for costs of this type of preventable hospitalization, which otherwise generates profit.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8491802/

Record 729

pmcid: PMC7985501

pmid: 33753828

doi: 10.1038/s41598-021-86029-0

title: Cardiothoracic imaging findings of Proteus syndrome.

authors

- Mirmomen SM
- Arai AE
- Turkbey EB
- Bradley AJ
- Sapp JC
- Biesecker LG
- Sirajuddin A

journal: Scientific reports

journalAbbr: Sci Rep

year: 2021

pubdate: 2021 Mar 22

volume: 11

issue: 1

pages: 6577

abstract: In this work, we sought to delineate the prevalence of cardiothoracic imaging findings of Proteus syndrome in a large cohort at our institution. Of 53 individuals with a confirmed diagnosis of Proteus syndrome at our institution from 10/2001 to 10/2019, 38 individuals (men, n = 23; average age = 24 years) underwent cardiothoracic imaging (routine chest CT, CT pulmonary angiography and/or cardiac MRI). All studies were retrospectively and independently reviewed by two fellowship-trained cardiothoracic readers. Disagreements were resolved by consensus. Differences between variables were analyzed via parametric and nonparametric tests based on the normality of the distribution. The cardiothoracic findings of Proteus syndrome were diverse, but several were much more common and included: scoliosis from bony overgrowth (94%), pulmonary venous dilation (62%), band-like areas of lung scarring (56%), and hyperlucent lung parenchyma (50%). In addition, of 20 individuals who underwent cardiac MRI, 9/20 (45%) had intramyocardial fat, mostly involving the endocardial surface of the left ventricular septal wall. There was no statistically significant difference among the functional cardiac parameters between individuals with and without intramyocardial fat. Only one individual with

intramyocardial fat had mildly decreased function (LVEF = 53%), while all others had normal ejection fraction.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7985501/

Record 730

pmcid: PMC8542968

pmid: 34707574

doi: 10.3389/fendo.2021.766253

title: Case Report: Lipoma of the Tuber Cinereum Mimicking a Pituitary Gland Abnormality in a Girl With Central Precocious Puberty.

authors

- Giacomozzi C
- Nicolì L
- Sozzi C
- Piovan E
- Maghnie M

journal: Frontiers in endocrinology

journalAbbr: Front Endocrinol (Lausanne)

year: 2021

pubdate: 2021

volume: 12

issue:

pages: 766253

abstract: INTRODUCTION: Magnetic Resonance Imaging (MRI) is the best approach to investigate the hypothalamic-pituitary region in children with central precocious puberty (CPP). Routine scanning is controversial in girls aged 6-8 year, due to the overwhelming prevalence of idiopathic forms and unrelated incidentalomas. Cerebral lipomas are rare and accidental findings, not usually expected in CPP. We report a girl with CPP and an unusually shaped posterior pituitary gland on SE-T1w sequences. CASE DESCRIPTION: A 7.3-year-old female was referred for breast development started at age 7. Her past medical history and physical examination were unremarkable, apart from the Tanner stage 2 breast. X-ray of the left-hand revealed a bone age 2-years ahead of her chronological age, projecting her adult height prognosis below the mid parental height. LHRH test and pelvic ultrasound were suggestive for CPP. Routine brain MRI sequences, SE T1w and TSE T2w, showed the posterior pituitary bright spot increased in size and stretched upward. The finding was considered as an anatomical variant, in an otherwise normal brain imaging. Patient was started on treatment with GnRH analogue. At a thorough revaluation, imaging overlap with adipose tissue was suspected and a new MRI scan with 3D-fat-suppression T1w-VIBE sequences demonstrated a lipoma of the tuber cinereum, bordering a perfectly normal neurohypophysis. 3D-T2w-SPACE sequences, acquired at first MRI scan, would have provided a more correct interpretation if rightly considered. CONCLUSION: This is the first evidence, to our knowledge, of a cerebral lipoma mimicking pituitary gland abnormalities. Our experience highlights the importance of considering suprasellar lipomas in the MRI investigation of children with CPP, despite their rarity, should the T1w sequences show an unexpected pituitary shape. 3D-T2w SPACE sequences could be integrated into standard ones, especially when performing MRI routinely, to avoid potential misinterpretations.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8542968/

Record 731

pmcid: PMC8586890

pmid: 33797780

doi: 10.1111/cen.14475

title: Central precocious puberty: Recent advances in understanding the aetiology and in the clinical approach.

authors

- Maione L
- Bouvattier C
- Kaiser UB

journal: Clinical endocrinology

journalAbbr: Clin Endocrinol (Oxf)

year: 2021

pubdate: 2021 Oct

volume: 95

issue: 4

pages: 542-555

abstract: Central precocious puberty (CPP) results from early activation of the hypothalamic-pituitary-gonadal (HPG) axis. The current state of knowledge of the complex neural network acting at the level of the hypothalamus and the GnRH neuron to control puberty onset has expanded, particularly in the context of molecular interactions. Along with these advances, the knowledge of pubertal physiology and pathophysiology has also increased. This review focuses on regulatory abnormalities occurring at the hypothalamic level of the HPG axis to cause CPP. The clinical approach to diagnosis of puberty and pubertal disorders is also reviewed, with a particular focus on aetiologies of CPP. The recent identification of mutations in MKRN3 and DLK1 in familial as well sporadic forms of CPP has changed the state of the art of the approach to patients with CPP. Genetic advances have also had important repercussions beyond consideration of puberty alone. Syndromic disorders and central nervous system lesions associated with CPP are also discussed. If untreated, these conditions may lead to adverse physical, psychosocial and medical outcomes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8586890/

Record 732

pmcid: PMC8637676

pmid: 34904029

doi: 10.4254/wjh.v13.i11.1552

title: Chelation therapy in liver diseases of childhood: Current status and response.

authors

- Seetharaman J
- Sarma MS

journal: World journal of hepatology

journalAbbr: World J Hepatol

year: 2021

pubdate: 2021 Nov 27

volume: 13

issue: 11

pages: 1552-1567

abstract: Chelation is the mainstay of therapy in certain pediatric liver diseases. Copper and iron related disorders require chelation. Wilson's disease (WD), one of the common causes of cirrhosis in children is treated

primarily with copper chelating agents like D-penicillamine and trientine. D-Penicillamine though widely used due its high efficacy in hepatic WD is fraught with frequent adverse effects resulting discontinuation. Trientine, an alternative drug has comparable efficacy in hepatic WD but has lower frequency of adverse effects. The role of ammonium tetra-thiomolybdate is presently experimental in hepatic WD. Indian childhood cirrhosis is related to excessive copper ingestion, rarely seen in present era. D-Penicillamine is effective in the early part of this disease with reversal of clinical status. Iron chelators are commonly used in secondary hemochromatosis of liver in hemolytic anemias. There are strict chelation protocols during bone marrow transplant. The role of iron chelation in neonatal hemochromatosis is presently not in vogue due to its poor efficacy and availability of other modalities of therapy. Hereditary hemochromatosis is rare in children and the use of iron chelators in this condition is limited.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8637676/

Record 733

pmcid: PMC8138374

pmid: 34041055

doi: 10.4103/jfmpc.jfmpc_1987_20

title: Clinical features, presentation and hormonal parameters in patients with pubertal gynecomastia.

authors

- Acharya SV

journal: Journal of family medicine and primary care

journalAbbr: J Family Med Prim Care

year: 2021

pubdate: 2021 Feb

volume: 10

issue: 2

pages: 648-651

abstract: OBJECTIVE: Gynecomastia is benign enlargement of breast in male. It is postulated that its development is primarily due to a mismatch in ratio of oestrogen and androgen at breast tissue. The aim of this study was to highlight the clinical features, presentation and hormonal parameters at the time of consultation. METHODS: All adolescent patients who attended endocrinology department with the complaint of breast enlargement were taken into study. We analysed their thyroid function test, oestrogen, testosterone, prolactin, alpha-fetoprotein, follicle-stimulating hormone, luteinizing hormone, beta HCG (human chorionic gonadotropin) and liver function. Clinical features and anthropometry were recorded. RESULTS: Out of 50 patients enrolled, 34 (68%) had bilateral gynaecomastia while 16 patients (32%) had unilateral disease. Mastalgia was seen in 44 patients (88%). Psychological disturbance in the form of depression was seen in 60% of adolescent boys with gynecomastia. Tanner stage B had positive correlation with oestrogen to testosterone (E2/TTE) ratio (r = 0.47; P = 0.034). All other hormonal parameters were normal. CONCLUSION: Mastalgia was one of the presenting complaints in majority of patients and many had bilateral enlargement. Altered oestrogen to testosterone ratio is altered in majority of patients thus may be reason for pubertal gynecomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8138374/

Record 734

pmcid: PMC8022032

pmid: 33867670

doi: 10.1297/cpe.30.99

title: Complete androgen insensitivity syndrome with accelerated onset of puberty due to a Sertoli cell tumor.

authors

- Izawa M
- Hisamatsu E
- Yoshino K
- Yoshida M
- Sato T
- Narumi S
- Hasegawa T
- Hamajima T

journal: Clinical pediatric endocrinology : case reports and clinical investigations : official journal of the Japanese Society for Pediatric Endocrinology

journalAbbr: Clin Pediatr Endocrinol

year: 2021

pubdate: 2021

volume: 30

issue: 2

pages: 99-104

abstract: Complete androgen insensitivity syndrome (CAIS) is caused by mutations in the androgen receptor gene. Patients with this syndrome have a 46,XY karyotype, male gonads, and normal female external genitalia. While the pre-pubertal risk of developing gonadal tumors is low in these patients, it increases with age. Most gonadal tumors arise from germ cells; stromal cell tumors are uncommon. Herein, we report a CAIS patient with a feminizing Sertoli cell tumor. The patient presented at 8 yr of age with breast enlargement and growth acceleration, concomitant with elevated serum estradiol levels and suppressed serum gonadotropin levels; these findings were inconsistent with CAIS. The patient underwent gonadectomy at 10 yr of age, and histology demonstrated presence of a non-malignant Sertoli cell tumor in the right gonad. We conclude that this is the first reported case of CAIS with accelerated onset of puberty resulting from a Sertoli cell tumor.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8022032/

Record 735

pmcid: PMC8219398

pmid: 34168861

doi: 10.1093/jscr/rjab276

title: Complications 18 years after polyacrylamide hydrogel augmentation mammoplasty: a case report and histopathological analysis.

authors

- DeLuca M
- Shapiro A
- Banayan E
- Zielinski G
- Karanetz I
- Asarian A
- Xiao P

journal: Journal of surgical case reports

journalAbbr: J Surg Case Rep

year: 2021

pubdate: 2021 Jun

volume: 2021

issue: 6

pages: rjab276

abstract: Polyacrylamide hydrogel (PAAG) is a synthetic substance previously used as an injectable material for augmentation mammoplasty. Current literature has demonstrated that the average time from PAAG injection to the onset of complication ranges from 6 to 39 months. We present a unique case report describing the onset of complications 18 years after PAAG augmentation mammoplasty. To the best of our knowledge, the presentation of a healthy female who experienced unprovoked expansion of breast tissue >15 years after polyacrylamide injection has not been previously reported in surgical literature. This suggests that serious complications of PAAG injection may occur later than the literature has previously described. Importantly, this case is the first demonstration of the successful surgical removal of polyacrylamide 18 years after injection. Additionally, this case also provides a histopathological analysis of breast capsules which showed evidence of an extensive chronic inflammatory reaction to polyacrylamide, consistent with previous reports.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8219398/

Record 736

pmcid: PMC8326153

pmid: 34367503

doi: 10.4330/wjc.v13.i7.183

title: Current concept in the diagnosis, treatment and rehabilitation of patients with congestive heart failure.

authors

- Sopek Merkaš I
- Slišković AM
- Lakušić N

journal: World journal of cardiology

journalAbbr: World J Cardiol

year: 2021

pubdate: 2021 Jul 26

volume: 13

issue: 7

pages: 183-203

abstract: Heart failure (HF) is a major public health problem with a prevalence of 1%-2% in developed countries. The underlying pathophysiology of HF is complex and as a clinical syndrome is characterized by various symptoms and signs. HF is classified according to left ventricular ejection fraction (LVEF) and falls into three groups: LVEF ≥ 50% - HF with preserved ejection fraction (HFpEF), LVEF < 40% - HF with reduced ejection fraction (HFrEF), LVEF 40%-49% - HF with mid-range ejection fraction. Diagnosing HF is primarily a clinical approach and it is based on anamnesis, physical examination, echocardiogram, radiological findings of the heart and lungs and laboratory tests, including a specific markers of HF - brain natriuretic peptide or N-terminal pro-B-type natriuretic peptide as well as other diagnostic tests in order to elucidate possible etiologies. Updated diagnostic algorithms for HFpEF have been recommended (H2FPEF, HFA-PEFF). New therapeutic options improve clinical outcomes as well as functional status in patients with HFrEF (e.g. , sodium-glucose cotransporter-2 - SGLT2 inhibitors) and such progress in treatment of HFrEF patients resulted in new working definition of the term "HF with recovered left ventricular ejection fraction". In line with rapid development of HF treatment, cardiac rehabilitation becomes an increasingly important part of overall approach to patients with

chronic HF for it has been proven that exercise training can relieve symptoms, improve exercise capacity and quality of life as well as reduce disability and hospitalization rates. We gave an overview of latest insights in HF diagnosis and treatment with special emphasize on the important role of cardiac rehabilitation in such patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8326153/

Record 737

pmcid: PMC8025724

pmid: 33870108

doi: 10.4274/ejbh.galenos.2020.6154

title: Cytological Evaluation of Pathological Male Breast Lesions.

authors

- Mondal K
- Mandal R

journal: European journal of breast health

journalAbbr: Eur J Breast Health

year: 2021

pubdate: 2021 Apr

volume: 17

issue: 2

pages: 103-111

abstract: OBJECTIVE: This study aimed to determine the cytodiagnostic spectrum of various male breast lesions, which were corroborated on histopathology as appropriate, to describe the process of the cytomorphology of some uncommon pathological lesions, and to discuss the reasons of their misdiagnoses. MATERIALS AND METHODS: In this 8-year study, a total of 114 patients underwent fine needle aspiration cytology (FNAC). In a representative case, nipple discharge from an 8-month-old child was examined. Confirmatory histopathology was obtained in 38 cases only. RESULTS: Gynecomastia was the most common (63.5%) male breast pathology. Invasive breast carcinoma of no special type was the most common variant of male breast malignancy. Half of the "gray zone" of cytological lesions was confirmed as cancer, but the rest were diagnosed as fibrocystic disease and intraductal papilloma. All cases with malignant cytology matched their corresponding histopathology. However, a tumor from an intraductal papillary carcinoma was miscued as ductal carcinoma on previous FNAC. CONCLUSION: Cytological evaluation of male breast lesions provides highly sensitive and specific results with excellent histologic reproducibility. Thus, it should be the ideal pretherapeutic diagnostic procedure for male breasts. However, some benign pathological conditions, which are particularly associated with epithelial hyperplasia, perplex the cytomorphologic scenario into the "gray zone."

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8025724/

Record 738

pmcid: PMC8213859

pmid: 33353864

doi: 10.1016/j.urolonc.2020.11.038

title: Decision regret, adverse outcomes, and treatment choice in men with localized prostate cancer: Results from a multi-site randomized trial.

authors

- Berry DL
- Hong F

- Blonquist TM
- Halpenny B
- Xiong N
- Filson CP
- Master VA
- Sanda MG
- Chang P
- Chien GW
- Jones RA
- Krupski TL
- Wolpin S
- Wilson L
- Hayes JH
- Trinh QD
- Sokoloff M

journal: Urologic oncology

journalAbbr: Urol Oncol

year: 2021

pubdate: 2021 Aug

volume: 39

issue: 8

pages: 493.e9-493.e15

abstract: INTRODUCTION: Men diagnosed with localized prostate cancer must navigate a highly preference-sensitive decision between treatment options with varying adverse outcome profiles. We evaluated whether use of a decision support tool previously shown to decrease decisional conflict also impacted the secondary outcome of post-treatment decision regret. METHODS: Participants were randomized to receive personalized decision support via the Personal Patient Profile-Prostate or usual care prior to a final treatment decision. Symptoms were measured just before randomization and 6 months later; decision regret was measured at 6 months along with records review to ascertain treatment choices. Regression modeling explored associations between baseline variables including race and D`Amico risk, study group, and 6-month variables regret, choice, and symptoms. RESULTS: At 6 months, 287 of 392 (73%) men returned questionnaires of which 257 (89%) had made a treatment choice. Of that group, 201 of 257 (78%) completely answered the regret scale. Regret was not significantly different between participants randomized to the P3P intervention compared to the control group (P = 0.360). In univariate analyses, we found that Black men, men with hormonal symptoms, and men with bowel symptoms reported significantly higher decision regret (all P < 0.01). Significant interactions were detected between race and study group (intervention vs. usual care) in the multivariable model; use of the Personal Patient Profile-Prostate was associated with significantly decreased decisional regret among Black men (P = 0.037). Interactions between regret, symptoms and treatment revealed that (1) men choosing definitive treatment and reporting no hormonal symptoms reported lower regret compared to all others; and (2) men choosing active surveillance and reporting bowel symptoms had higher regret compared to all others. CONCLUSION: The Personal Patient Profile-Prostate decision support tool may be most beneficial in minimizing decisional regret for Black men considering treatment options for newly-diagnosed prostate cancer. TRIAL REGISTRATION: NCT01844999.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8213859/

Record 739

pmcid: PMC8683245

pmid: 34925926

doi: 10.1155/2021/2999349

title: Delayed Diagnosis of McCune-Albright Syndrome.

authors

- Fantahun B
- Desta S

journal: Case reports in genetics

journalAbbr: Case Rep Genet

year: 2021

pubdate: 2021

volume: 2021

issue:

pages: 2999349

abstract: BACKGROUND: McCune-Albright syndrome (MAS) is a rare heterogeneous genetic disorder that is characterized by a triad of polyostotic fibrous dysplasia (FD), café au lait spots (CAL), and multiple hyperfunctional endocrinopathies. In general, it is diagnosed clinically. From the triads, two of the findings are enough to make the diagnosis, but genetic testing can be done if it is available. Case Presentation . We report a female child who was symptomatic since the neonatal period with skin hyperpigmentation, breast enlargement, and vaginal bleeding. She was diagnosed with MAS at the age of five years. She had pathological fractures at multiple sites and had raised thyroid hormones since the age of 3½ years. The child developed severe morbidity as the result of delayed diagnosis and currently became wheelchair dependent. CONCLUSION: Thorough patient evaluation and appropriate interpretation of findings are crucial steps for timely diagnosis of MAS and better patient care outcomes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8683245/

Record 740

pmcid: PMC8102217

pmid: 33968703

doi: 10.21037/gs-20-854

title: Delayed unilateral hematoma after reconstructive and aesthetic breast surgery with implants in Asian patients: two case reports.

authors

- Lee JH
- Hong HK
- Kim WH
- Kim HJ
- Lee J
- Park HY
- Yang JD
- Lee JS

journal: Gland surgery

journalAbbr: Gland Surg

year: 2021

pubdate: 2021 Apr

volume: 10

issue: 4

pages: 1515-1522

abstract: Hematomas represent one of the postoperative complications in patients undergoing reconstructive or aesthetic breast surgery with a silicone implant. Although there are few reports of intracapsular hematoma, those presenting late hematoma after reconstructive and aesthetic augmentation surgeries are rarer. This study reported two Asian patients with late hematoma after reconstruction and aesthetic breast surgery. A 54-year-old female patient underwent bilateral nipple-sparing mastectomy with immediate breast reconstruction using anatomically shaped textured implant for intraductal carcinoma in August 2019. Contralateral nipple-sparing mastectomy was performed for the BRCA gene mutation on the left breast, which was immediately reconstructed with an anatomically shaped textured implant. In a 1-year postoperative magnetic resonance imaging evaluation, an extracapsular hematoma was found on the right side, which was removed following the removal of both implants. Another case was a 63-year-old female patient who underwent augmentation of both breasts with smooth round implants and experienced right unilateral swelling and painless firmness about 30 years postoperatively. A preoperative magnetic resonance imaging evaluation showed both intracapsular and extracapsular ruptures on the right breast and a bulging implant herniation on the left breast. During the operation, hematoma, implants, and capsule were all removed. The excised capsule was sent for histological evaluation. Slightly dark colored blood was emptied before removing the semisolid-state intracapsular hematoma. In both cases, the patients responded well postoperatively and were discharged to their homes with no postsurgical complications, including seroma, or additional hematoma on the breasts. The etiology of late hematoma following breast augmentation or reconstruction has been poorly characterized. Further reports are needed to clearly establish the reasons for this increase in late hematoma formation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8102217/

Record 741

pmcid: PMC8510514

pmid: 34691199

doi: 10.7573/dic.2021-8-6

title: Dermatology: how to manage acne vulgaris.

authors

- Leung AK
- Barankin B
- Lam JM
- Leong KF
- Hon KL

journal: Drugs in context

journalAbbr: Drugs Context

year: 2021

pubdate: 2021

volume: 10

issue:

pages:

abstract: BACKGROUND: Acne vulgaris is the most common skin disease that can lead to disfigurement and psychological distress. This article aims to provide a narrative updated review on the management of acne vulgaris. METHODS: A PubMed search was performed with Clinical Queries using the key term "acne". The

search strategy included clinical trials, meta-analyses, randomized controlled trials, observational studies and reviews. The search was restricted to articles published in English. RESULTS: Treatments of acne include proper skin care, topical medications, oral medications and procedural therapies. Topical agents are the first-line treatment for mild-to-moderate acne and can be used as combination therapy for more severe acne. Systemic therapies are usually prescribed for the initial treatment of moderate-to-severe acne as well as for acne that is refractory to topical therapies. CONCLUSION: Topical retinoids are the drugs of choice for the treatment and maintenance therapy of patients with mild-to-moderate acne vulgaris. Depending on the severity of the acne, topical retinoids may be used alone or in combination with benzoyl peroxide and topical or oral antibiotics. Oral antibiotics are an important therapy for inflammatory acne unresponsive to topical therapy. Neither topical nor oral antibiotics should be used as monotherapy. Oral contraceptives and/or spironolactone are useful for many women with acne. Oral isotretinoin is the drug of choice for severe, extensive, nodular acne vulgaris but is also often used in moderate cases where scarring is evident, acne-related psychosocial distress is significant or other treatment modalities have failed.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8510514/

Record 742

pmcid: PMC8530913

pmid: 34314814

doi: 10.1016/j.ijrobp.2021.07.1692

title: Development and Pilot Implementation of a Remote Monitoring System for Acute Toxicity Using Electronic Patient-Reported Outcomes for Patients Undergoing Radiation Therapy for Breast Cancer.

authors

- Lapen K
- Sabol C
- Tin AL
- Lynch K
- Kassa A
- Mabli X
- Ford J
- Cha E
- Bernstein MB
- Braunstein LZ
- Cahlon O
- Daly BM
- Sandler K
- McCloskey SA
- Vickers AJ
- Khan AJ
- Gillespie EF

journal: International journal of radiation oncology, biology, physics

journalAbbr: Int J Radiat Oncol Biol Phys

year: 2021

pubdate: 2021 Nov 15

volume: 111

issue: 4

pages: 979-991

abstract: PURPOSE: We aimed to develop and study the implementation of a remote system for toxicity assessment and management of acute side effects of breast radiation using electronic patient-reported outcomes (ePROs). METHODS AND MATERIALS: A response-adapted Patient-Reported Outcomes Common Terminology Criteria for Adverse Events-based assessment for breast radiation toxicity was administered weekly during and for 8 weeks after radiation from June 2019 to July 2020. The care team received alerts when "severe" symptoms were reported by patients, who were then contacted. Treatment, clinic, and sociodemographic characteristics were abstracted from patient records. A subsample of patients and care team members was qualitatively interviewed at follow-up. RESULTS: Overall, 5787 assessments were sent to 678 patients, of whom 489 (72%) completed 2607 assessments (45%). Moderate or greater toxicity was reported by 419 responders (86%; 95% CI, 82%-89%). Clinician alerts for severe toxicity were generated for 264 assessments among 139 unique patients, of which 83% occurred posttreatment. The proportion of surveys that prompted an alert was significantly higher after treatment (219 [13%]) than during treatment (45 [5%]) (P < .001). Survey completion rates in the posttreatment period were higher among patients undergoing partial breast irradiation than postmastectomy radiation (incidence rate ratio, 0.70; 95% CI, 0.60-0.81) (P < .001) despite these patients experiencing less severe toxicity. Interviews (15) found that patients had a positive experience with ePROs, although many thought the primary purpose was for research rather than symptom management. CONCLUSIONS: With the majority of toxicity occurring after breast radiation has ended, remote symptom monitoring with ePROs appears to fill a gap in clinical practice, particularly for patients undergoing shorter courses of radiation. It is important to properly onboard patients and explain that the purpose of ePROs is to aid clinical care. Further research is needed to determine whether the costs associated with ePROs can be offset by reducing routine clinic visits and whether this approach is acceptable and appropriate.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8530913/

Record 743

pmcid: PMC8442957

pmid: 34539579

doi: 10.3389/fendo.2021.735875

title: Diagnostic Accuracy of Female Pelvic Ultrasonography in Differentiating Precocious Puberty From Premature Thelarche: A Systematic Review and Meta-analysis.

authors

- Nguyen NN
- Huynh LBP
- Do MD
- Yang TY
- Tsai MC
- Chen YC

journal: Frontiers in endocrinology

journalAbbr: Front Endocrinol (Lausanne)

year: 2021

pubdate: 2021

volume: 12

issue:

pages: 735875

abstract: BACKGROUND: The gonadotropin-releasing hormone (GnRH) stimulation test is the benchmark for diagnosing precocious puberty (PP). However, it is invasive, time-consuming, costly, and may create an unpleasant experience for participants. Moreover, some overlaps may occur between PP and premature

thelarche (PT) in the early stage of PP. Female pelvic ultrasonography may provide additional information to help differentiate PP from PT and subsequently initiate early treatment. In this study, we aimed to first directly compare pelvic ultrasonography parameters between PP and PT groups and secondly, investigate their diagnostic accuracy compared with the GnRH stimulation test. METHODS: A systematic search of the PubMed/MEDLINE, EMBASE, Scopus, and Cochrane Library databases was performed up to March 31, 2021. All types of studies, except for case reports and review articles, were included. The GnRH stimulation test was used to confirm PP diagnosis. Those whose organic conditions might cause PP were excluded. The mean, standard deviation, sensitivity, and specificity of each parameter were documented. Forest plots were constructed to display the estimated standardized mean differences (SMDs) from each included study and the overall calculations. A bivariate model was used to calculate the pooled sensitivity, specificity, positive likelihood ratio (PLR), negative likelihood ratio (NLR), and diagnostic odds ratio (DOR). RESULTS: A total of 13 studies were included for analysis. The SMDs (95% confidence interval - CI) in ovarian volume, fundal-cervical ratio, uterine length, uterine cross-sectional area, and uterine volume between PP and PT groups were 1.12 (0.78-1.45; $p < 0.01$), 0.90 (0.07-1.73; $p = 0.03$), 1.38 (0.99-1.78; $p < 0.01$), 1.06 (0.61-1.50; $p < 0.01$), and 1.21 (0.84-1.58; $p < 0.01$), respectively. A uterine length of 3.20 cm yielded a pooled sensitivity of 81.8% (95% CI 78.3%-84.9%), specificity of 82.0% (95% CI 61.0%-93.0%), PLR of 4.56 (95% CI 2.15-9.69), NLR of 0.26 (95% CI 0.17-0.39), and DOR of 19.62 (95% CI 6.45-59.68). The area under the summary receiver operating characteristics curve was 0.82. CONCLUSION: Female pelvic ultrasonography may serve as a complementary tool to the GnRH stimulation test in differentiating PP from PT. SYSTEMATIC REVIEW REGISTRATION: https://www.crd.york.ac.uk/prospero/display_record.php?ID=CRD42021232427, ID: CRD42021232427.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8442957/

Record 744

pmcid: PMC8663041

pmid:

doi: 10.1017/mdh.2021.17

title: Doctors, families and the industry in the clinic: the management of ‘intersex’ children in Swiss paediatric medicine (1945–1970).

authors

- Janett M
- Althaus A
- Hulverscheidt M
- Gobet R
- Streuli J
- Condrau F

journal: Medical History

journalAbbr: Med Hist

year: 2021

pubdate: 2021 Jul

volume: 65

issue: 3

pages: 286-305

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8663041/

Record 745

pmcid: PMC8516320

pmid: 34667677

doi: 10.7759/cureus.18001

title: Does Male Infantile Galactocele Always Necessitate Surgical Intervention?

authors

- Algethami NE
- Taha A
- Altalhi WA
- Alsulaimani AI

journal: Cureus

journalAbbr: Cureus

year: 2021

pubdate: 2021 Sep

volume: 13

issue: 9

pages: e18001

abstract: Galactocele is cystic dilatation of the mammary gland, which contains milk. Galactocele is usually presented as a painless breast enlargement with a fluctuant, soft, mobile, and non-tender mass in the breast. It is a rare disorder in the male pediatric age group, with only 31 cases reported in the literature. In this article, we present another case of unilateral galactocele in a 14-month-old male infant. Ultrasound imaging confirmed a simple cystic hypoechoic mass in the right breast, and needle aspiration was performed before surgery as a diagnostic procedure (a 1-cc extract revealed a milky-colored fluid).

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8516320/

Record 746

pmcid: PMC8481221

pmid: 34145475

doi: 10.1007/s00266-021-02351-y

title: Double-Unit Superomedio-Central (DUS) Pedicle Inverted-T Reduction Mammaplasty in Gigantomastia: A 7-year Single-Center Retrospective Study.

authors

- Wolter A
- Fertsch S
- Munder B
- Stambera P
- Schulz T
- Hagouan M
- Janku D
- Staemmler K
- Grueter L
- Abu-Abdallah N
- Becker K
- Aufmesser B

- Kornetka J
- Andree C

journal: Aesthetic plastic surgery

journalAbbr: Aesthetic Plast Surg

year: 2021

pubdate: 2021 Oct

volume: 45

issue: 5

pages: 2061-2074

abstract: INTRODUCTION: Reduction mammaplasty in patients with gigantomastia is challenging. The Double-Unit technique with a Superomedio-Central pedicle and inverted-T incision is the standard technique for reduction mammaplasty in our clinic. The aim of this study was to review our approach in cases with gigantomastia in comparison with the current literature. PATIENTS AND METHODS: From 01/2011 to 12/2017, we performed 831 reduction mammaplasties in 630 patients. The Double-Unit Superomedio-Central (DUS) pedicle and inverted-T incision was implemented as a standard procedure for gigantomastia. Patient demographics and the outcome parameters complication rate, patient satisfaction with the aesthetic result, nipple sensibility, and surgical revision rate were obtained and retrospectively analyzed. RESULTS: In 37 patients, 55 reduction mammaplasties were performed with more than 1000 g per breast. Mean resection weight was 1311 g on right side and 1289 g on left side. Mean age was 52.5 years, mean body mass index was 32.8 kg/m 2 , mean sternal-notch-to-nipple distance was 38.3 cm. A free NAC graft was necessary in four breasts. Overall complication rate was 14.5%; secondary surgical revision rate was 12.7%. 91% of the patients were "very satisfied" and "satisfied" with the aesthetic result. Nipple sensibility was rated "high" and "medium" in 83%. CONCLUSION: The Double-Unit technique with a Superomedio-Central pedicle and inverted-T incision is very effective to achieve volume reduction and aesthetically pleasing reproducible results with a low complication rate in cases with gigantomastia. LEVEL OF EVIDENCE: Level of Evidence This journal requires that authors assign a level of evidence to each article. For a full description of these Evidence-Based Medicine ratings, please refer to the Table of Contents or the online Instructions to Authors www.springer.com/00266 .

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8481221/

Record 747

pmcid: PMC8395071

pmid: 34445068

doi: 10.3390/ijms22168360

title: Effectiveness Assessment of a Modified Preservation Solution Containing Thyrotropin or Follitropin Based on Biochemical Analysis in Perfundates and Homogenates of Isolated Porcine Kidneys after Static Cold Storage.

authors

- Ostróżka-Cieślik A
- Dolińska B
- Ryszka F

journal: International journal of molecular sciences

journalAbbr: Int J Mol Sci

year: 2021

pubdate: 2021 Aug 4

volume: 22

issue: 16

pages:

abstract: In this paper, we assess the nephroprotective effects of thyrotropin and follitropin during ischaemia. The studies were performed in vitro in a model of isolated porcine kidneys stored in Biolasol (FZNP, Biochefa, Sosnowiec, Poland) and modified Biolasol (TSH: 1 µg/L; FSH 1 µg/L). We used the static cold storage method. The study was carried out based on 30 kidneys. The kidneys were placed in 500 mL of preservation solution chilled to 4 °C. The samples for biochemical tests were collected during the first kidney perfusion (after 2 h of storage) and during the second perfusion (after 48 h of storage). The results of ALT, AST, and LDH activities confirm the effectiveness of Biolasol + p-TSH in maintaining the structural integrity of renal cell membranes. Significantly reduced biochemical parameters of kidney function, i.e., creatinine and protein concentrations were also observed after 48 h storage. The protective effect of Biasol + p-TSH is most pronounced after 2 h of storage, suggesting a mild course of damage thereafter. A mild deterioration of renal function was observed after 48 h. The results of our analyses did not show any protective effect of Biolasol + p-FSH on the kidneys during ischaemia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8395071/

Record 748

pmcid: PMC8687027

pmid: 35018050

doi: 10.4103/jpbs.jpbs_391_21

title: Efficacy of Spironolactone in Adult Acne in Polycystic Ovary Syndrome Patients an Original Research.

authors

- Rani N
- Kumar P
- Mishra AK
- Sankuratri BYV
- Sethi S
- Gelada K
- Tiwari H

journal: Journal of pharmacy & bioallied sciences

journalAbbr: J Pharm Bioallied Sci

year: 2021

pubdate: 2021 Nov

volume: 13

issue: Suppl 2

pages: S1659-S1663

abstract: INTRODUCTION: This prospective clinical trial was designed to assess the effects of a long-term therapy with spironolactone, with and without dietary-induced weight-loss, on clinical features, lipid profile, and insulin levels in women with polycystic ovary syndrome (PCOS). MATERIALS AND METHODS: Twenty-five patients (range of age 16-32 year; 13 lean and 12 overweight) fulfilling formal diagnostic criteria for PCOS (oligomenorrhea and/or amenorrhea, biochemical and/or clinical evidence of hyperadrogenism) were studied at baseline and then received oral spironolactone (100 mg/die) for 12 months; association with lifestyle modifications was recommended to all overweight patients. Clinical, endocrine, and metabolic parameters (oral glucose tolerance test [OGTT], lipid profile) were measured at baseline and at the end of the antiandrogen treatment. RESULTS: The therapy was associated with a significant average decline of triglycerides in overweight subjects and with increased high-density lipoprotein-cholesterol levels in lean patients. The insulin levels at 60 min during OGTT, homeostasis model assessment-insulin resistance and area under curve of insulin were significantly lowered in overweight women after 12 months of spironolactone and weight loss and

no negative changes in insulin secretion and sensitivity were observed in PCOS women after pharmacological treatment alone. CONCLUSION: The efficacy of spironolactone on the androgenic clinical aspects of PCOS has been confirmed in this study. Furthermore, our data show that long-term treatment with spironolactone exerts no negative effects on lipoprotein profile and glucose metabolism; more relevant beneficial effects on glucose and lipid metabolism were observed when the antiandrogen was associated with weight loss in overweight PCOS women.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8687027/

Record 749

pmcid: PMC8144348

pmid: 34041454

doi: 10.1016/j.isci.2021.102414

title: Estradiol induces allosteric coupling and partitioning of sex-hormone-binding globulin monomers among conformational states.

authors

- Jasuja R
- Spencer D
- Jayaraj A
- Peng L
- Krishna M
- Lawney B
- Patel P
- Jayaram B
- Thayer KM
- Beveridge DL
- Bhasin S

journal: iScience

journalAbbr: iScience

year: 2021

pubdate: 2021 Jun 25

volume: 24

issue: 6

pages: 102414

abstract: Sex-hormone-binding globulin (SHBG) regulates the transport and bioavailability of estradiol. The dynamics of estradiol's binding to SHBG are incompletely understood, although it is believed that estradiol binds to each monomer of SHBG dimer with identical affinity (K d ~2 nM). Contrary to the prevalent view, we show that estradiol's binding to SHBG is nonlinear, and the "apparent" K d changes with varying estradiol and SHBG concentrations. Estradiol's binding to each SHBG monomer influences residues in the ligand-binding pocket of both monomers and differentially alters the conformational and energy landscapes of both monomers. Monomers are not energetically or conformationally equivalent even in fully bound state. Estradiol's binding to SHBG involves bidirectional, inter-monomeric allostery that changes the distribution of both monomers among various energy and conformational states. Inter-monomeric allostery offers a mechanism to extend the binding range of SHBG and regulate hormone bioavailability as estradiol concentrations vary widely during life.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8144348/

Record 750

pmcid: PMC7863556

pmid: 33522308

doi: 10.1177/0963689721991477

title: Estrogen and Androgen Receptor Inhibitors: Unexpected Allies in the Fight Against COVID-19.

authors

- Bravaccini S
- Fonzi E
- Tebaldi M
- Angeli D
- Martinelli G
- Nicolini F
- Parrella P
- Mazza M

journal: Cell transplantation

journalAbbr: Cell Transplant

year: 2021

pubdate: 2021 Jan-Dec

volume: 30

issue:

pages: 963689721991477

abstract: TRANSLATIONAL RELEVANCE: No prophylactic treatments for COVID-19 have been clearly proven and found. In this pandemic context, cancer patients constitute a particularly fragile population that would benefit the best from such treatments, a present unmet need. TMPRSS2 is essential for COVID-19 replication cycle and it is under androgen control. Estrogen and androgen receptor dependent cues converge on TMPRSS2 regulation through different mechanisms of action that can be blocked by the use of hormonal therapies. We believe that there is enough body of evidence to foresee a prophylactic use of hormonal therapies against COVID-19 and this hypothesis can be easily tested on cohorts of breast and prostate cancer patients who follow those regimens. In case of pandemic, if the protective effect of hormonal therapies will be proven on cancer patients, the use of specific hormonal therapies could be extended to other oncological groups and to healthy individuals to decrease the overall risk of infection by SARS-CoV-2. UNLABELLED: Given the COVID-19 coronavirus emergency, a special focus is needed on the impact of this rapidly spreading viral infection on cancer patients. Androgen receptor (AR) signaling in the transmembrane protease serine 2 (TMPRSS2) regulation is emerging as an important determinant of severe acute respiratory syndrome coronavirus 2 (SARS-CoV-2) susceptibility. In our study, we analyzed AR and TMPRSS2 expression in 17,352 normal and 9,556 cancer tissues from public repositories and stratified data according to sex and age. The emerging picture is that some patient groups may be particularly susceptible to SARS-CoV-2 infection and may benefit from antiandrogen- or tamoxifen-based therapies. These findings are relevant to choose proper treatments in order to protect cancer patients from concomitant SARS-CoV-2 contagion and related symptoms and put forward the idea that hormonal therapies could be used as prophylactic agents against COVID-19.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7863556/

Record 751

pmcid: PMC7983493

pmid: 33845455

doi: 10.1530/EDM-20-0214

title: Estrogen-secreting adrenocortical tumor in a postmenopausal woman: a challenging diagnosis.

authors

- Mermejo LM
- Carvalho FG
- Molina CAF
- Tucci S
- Muglia VF
- Elias J
- Elias PCL
- Castro M
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journal: Endocrinology, diabetes & metabolism case reports

journalAbbr: Endocrinol Diabetes Metab Case Rep

year: 2021

pubdate: 2021 Jan 1

volume: 2021

issue:

pages:

abstract: SUMMARY: Adrenocortical carcinoma (ACC) is a malignant disorder with rapid evolution and severe prognosis in adults and most produce cortisol and androgen. Estrogen-secreting adrenocortical carcinomas are extremely rare, especially in women, tend to be larger and have worse prognosis compared with other types of ACCs. We report the case of a 58-year-old woman who presented with bilateral breast enlargement and postmenopausal genital bleeding. She presented high estradiol (818 pg/mL - 25 times above upper normal limit for postmenopausal women) and testosterone (158 ng/dL - 2 times above upper normal limit) levels and no suppression of cortisol after overnight 1 mg dexamethasone test (12.5 µg/dL; normal reference value: < 1.8 µg/dL). The patient had no clinical features of cortisol excess. MRI showed a 12 cm tumor in the right adrenal. Clinical findings of bilateral breast enlargement and postmenopausal genital bleeding with no signs of hypercortisolism associated with hormonal findings of elevated estradiol and testosterone levels would indicate either an ovarian etiology or an adrenal etiology; however, in the context of plasma cortisol levels non-suppressive after dexamethasone test and the confirmation of an adrenal tumor by MRI, the diagnosis of an adrenal tumor with mixed hormonal secretion was made. The patient underwent an open right adrenalectomy and pathological examination revealed an ACC with a Weiss' score of 6. Estradiol and testosterone levels decreased to normal range soon after surgery. She was put on mitotane treatment as adjuvant therapy, but due to side effects, we were unable to up-titrate the dose and she never achieved serum mitotane dosage above the desired 14 µg/mL. The patient remained in good health without any local recurrence or metastasis until 5 years after surgery, when increased levels of estradiol (81 pg/mL - 2.5 times above upper normal limit) and testosterone (170 ng/dL - 2.1 times above upper normal limit) were detected. MRI revealed a retroperitoneal nodule measuring 1.8 × 1.2 cm. The pathological finding confirmed the recurrence of the estrogen-secreting ACC with a Weiss' score of 6. After the second procedure, patient achieved normal estrogen and androgen serum levels and since then she has been followed for 3 years. The overall survival was 8 years after the diagnosis. In conclusion, although extremely rare, a diagnosis of an estrogen-secreting ACC should be considered as an etiology in postmenopausal women presenting with bilateral breast enlargement, genital bleeding and increased pure or prevailing estrogen secretion. LEARNING POINTS: Estrogen-secreting adrenocortical carcinomas are exceedingly rare in adults and account for 1-2% of adrenocortical carcinomas. Estrogen-secreting adrenal tumors can be present in females, but are even more rare, we found few cases described in the literature. In women, they present with precocious puberty or postmenopausal bleeding. Feminization in the context of an adrenal tumor is considered almost pathognomonic of malignancy. Feminizing ACCs tend to be larger and with worse prognosis compared with nonfeminizing ACCs.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7983493/

Record 752

pmcid: PMC8025730

pmid: 33870116

doi: 10.4274/ejbh.galenos.2021.6336

title: Evaluation of Factors Related to Postoperative Complications in Patients Who Underwent Reduction Mammoplasty.

authors

- Toplu G
- Altınel D
- Serin M

journal: European journal of breast health

journalAbbr: Eur J Breast Health

year: 2021

pubdate: 2021 Apr

volume: 17

issue: 2

pages: 157-164

abstract: OBJECTIVE: This study aimed to investigate whether there was an increase in the number of postoperative complications in patients undergoing reduction mammoplasty depending on the technique used (i.e., pedicle type, skin incision pattern), existence of concomitant diseases, and presence of other risk factors. MATERIALS AND METHODS: A total of 186 patients who underwent breast reduction between 2013 and 2018 (bilateral, n = 170; unilateral, n = 16) were included in the study. A retrospective review of the data of patients who underwent reduction mammoplasty, which was performed by the same surgical team in a single institution over a 6-year period, was carried out. Superomedial, superior, and inferior pedicles were used in 99, 55, and 32 patients, respectively. The median follow-up period was 4 years. RESULTS: The median patient age was 45 (range: 16-75) years. The median total reduction weight was 2,194 (range: 80-4,800) grams. The median distance between the sternal notch and nipple was 31 cm (range: 24-45 cm) for the right breast and 30 cm (range: 22-45 cm) for the left breast. The overall complication rate was 6.9%. The complication rates in patients with and without any concomitant diseases were 10.2% and 4.6%, respectively. The overall complication rate was significantly higher in patients with smoking habit, accessory breasts, progesterone use, cerebrovascular disease, morbid obesity (Body Mass Index ≥40 kg/m 2), and thalassemia. CONCLUSION: Our analysis shows that the presence of concomitant diseases increases the risk for postoperative complications in patients who underwent reduction mammoplasty. Our findings do not suggest that any of the techniques have significant superiority to each other in terms of pedicle safety and overall complication rate.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8025730/

Record 753

pmcid: PMC7828439

pmid: 33450918

doi: 10.3390/ijms22020724

title: Evaluation of the Multimycotoxin-Degrading Efficiency of Rhodococcus erythropolis NI1 Strain with the Three-Step Zebrafish Microinjection Method.

authors

- Garai E
- Risa A

- Varga E
- Cserháti M
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journal: International journal of molecular sciences

journalAbbr: Int J Mol Sci

year: 2021

pubdate: 2021 Jan 13

volume: 22

issue: 2

pages:

abstract: The multimycotoxin-degrading efficiency of the Rhodococcus erythropolis NI1 strain was investigated with a previously developed three-step method. NI1 bacterial metabolites, single and combined mycotoxins and their NI1 degradation products, were injected into one cell stage zebrafish embryos in the same doses. Toxic and interaction effects were supplemented with UHPLC-MS/MS measurement of toxin concentrations. Results showed that the NI1 strain was able to degrade mycotoxins and their mixtures in different proportions, where a higher ratio of mycotoxins were reduced in combination than single ones. The NI1 strain reduced the toxic effects of mycotoxins and mixtures, except for the AFB1+T-2 mixture. Degradation products of the AFB1+T-2 mixture by the NI1 strain were more toxic than the initial AFB1+T-2 mixture, while the analytical results showed very high degradation, which means that the NI1 strain degraded this mixture to toxic degradation products. The NI1 strain was able to detoxify the AFB1, ZEN, T-2 toxins and mixtures (except for AFB1+T-2 mixture) during the degradation experiments, which means that the NI1 strain degraded these to non-toxic degradation products. The results demonstrate that single exposures of mycotoxins were very toxic. The combined exposure of mycotoxins had synergistic effects, except for ZEN+T-2 and AFB1+ZEN +T-2, whose mixtures had very strong antagonistic effects.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7828439/

Record 754

pmcid: PMC8686322

pmid: 33167841

doi: 10.2174/1570159X18666201109090824

title: Expanding the Arsenal Against Huntington's Disease-Herbal Drugs and Their Nanoformulations.

authors

- Vishwas S
- Gulati M
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- Singh SK
- Awasthi A
- Khan A
- Goyal A
- Bansal A
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- Arora S
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journal: Current neuropharmacology

journalAbbr: Curr Neuropharmacol

year: 2021

pubdate: 2021

volume: 19

issue: 7

pages: 957-989

abstract: Huntington's disease (HD) is an autosomal fatal genetic disease in which degeneration of neuronal cells occurs in the central nervous system (CNS). Commonly used therapeutics are cludemonoamine depletors, antipsychotics, antidepressants, and tranquilizers. However, these drugs cannot prevent the psychotic, cognitive, and behavioral dysfunctions associated with HD. In addition to this, their chronic use is limited by their long-term side effects. Herbal drugs offer a plausible alternative to this and have shown substantial therapeutic effects against HD. Moreover, their safety profile is better in terms of side effects. However, due to limited drug solubility and permeability to reach the target site, herbal drugs have not been able to reach the stage of clinical exploration. In recent years, the paradigm of research has been shifted towards the development of herbal drugs based nanoformulations that can enhance their bioavailability and blood-brain barrier permeability. The present review covers the pathophysiology of HD, available biomarkers, phytomedicines explored against HD, ongoing clinical trials on herbal drugs exclusively for treating HD and their nanocarriers, along with their potential neuroprotective effects.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8686322/

Record 755

pmcid: PMC7783980

pmid: 33402108

doi: 10.1186/s12884-020-03504-8

title: Exploring the experiences related to postpartum changes: perspectives of mothers and healthcare providers in Iran.

authors

- Asadi M
- Noroozi M
- Alavi M

journal: BMC pregnancy and childbirth

journalAbbr: BMC Pregnancy Childbirth

year: 2021

pubdate: 2021 Jan 5

volume: 21

issue: 1

pages: 7

abstract: BACKGROUND: Numerous changes occur in different aspects of women's lives in the postpartum period. Women's adjusting with problems and taking advantage of this opportunity can develop their personality.

In this regard, accurate knowledge of their experiences and feelings is necessary to help them to benefit from this period. Therefore, the present study aimed to explore the experiences related to postpartum changes in women. METHODS: In the present qualitative study, 23 participants, including women of childbearing age who gave birth and healthcare providers (midwives and obstetricians) in Isfahan, Iran were selected using purposive sampling with a maximum variation strategy. Data were collected through in-depth semi structured interviews, field notes, and daily notes, and simultaneously analyzed using the conventional qualitative content analysis. RESULTS: The data analysis results led to the extraction of three main categories including "feeling of decreased female attractiveness" (with two sub-categories of " feeling of decreased beauty" and "feeling of decreased sexual function"), "feeling of insolvency and helplessness" (with two sub-categories of "physical burnout", and "mental preoccupations") and "beginning a new period in life" (with three sub-categories of "changing the meaning of life", "feeling of maturity" and "deepening the communication"). CONCLUSIONS: Findings of this study can provide a good context for designing interventions to improve the women's quality of life by explaining and highlighting their experiences in the postpartum period. In this regard, providing sufficient empathy, social and psychological support from family members (especially husband), performing appropriate educational interventions and also regular assessment of women's psychological state by healthcare providers in postpartum period can reduce their concerns and help to improve their health.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7783980/

Record 756

pmcid: PMC8471943

pmid: 34574781

doi: 10.3390/ijerph18189854

title: Exploring Trans People's Narratives of Transition: Negotiation of Gendered Bodies in Physical Activity and Sport.

authors

- Pereira-García S
- Devís-Devís J
- López-Cañada E
- Fuentes-Miguel J
- Sparkes AC
- Pérez-Samaniego V

journal: International journal of environmental research and public health

journalAbbr: Int J Environ Res Public Health

year: 2021

pubdate: 2021 Sep 18

volume: 18

issue: 18

pages:

abstract: This paper explores how trans people who make transitions negotiate their gendered bodies in different moments of this process, and how their narrative storylines are emplotted in physical activity and (non)organized sports (PAS) participation. A qualitative semi-structured interview-based study was developed to analyze the stories of eight trans people (three trans women, two trans men, and three nonbinary persons) who participated in PAS before and during their gender disclosure. A thematic analysis was conducted to identify the patterns in the transition process and the structural analysis of the stories from the interviews. Three transition moments (the closet, opening up, and reassuring) were identified from the thematic analysis. Most participants showed difficulties in achieving their PAS participation during the two earlier moments. The predominance of failure storylines was found particularly in men, while success was more likely to appear in women because their bodies and choices fitted better with their PAS gender ideals. The nonbinary trans persons present alternative

storylines in which corporeality has less influence on their PAS experiences. The knowledge provided on the moments and the stories of transition help to explain trans people's (non)involvement in PAS and to guide policymaking and professional action in PAS fields.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8471943/

Record 757

pmcid: PMC8769100

pmid: 35070603

doi: 10.1097/GOX.0000000000003960

title: Extreme Gigantomastia Caused by Pseudoangiomatous Stromal Hyperplasia at Hopital Universitaire de Mirebalais: A Case Report.

authors

- Jean-Louis WF
- Bowder A
- Dupont CR
- Delva R
- Jean-Louis M
- Mixter RC
- Duffy FJ

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2021

pubdate: 2021 Dec

volume: 9

issue: 12

pages: e3960

abstract: Gigantomastia is a rare pathologic condition characterized by an excessive and abnormal breast development. To date, there is no universally accepted definition or classification of this disease. The purpose of this article is to present a case of extreme and disabling gigantomastia caused by pseudoangiomatous stromal hyperplasia, the rarest etiology of gigantomastia. Very few cases have been listed in the literature. We believe this case of gigantomastia is unique because of its associated severe morbidity considerably impacting the patient's life. Good aesthetic results and high patient satisfaction can be achieved with bilateral breast reduction and free nipple areola grafts.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8769100/

Record 758

pmcid: PMC8690961

pmid: 34930443

doi: 10.1186/s13256-021-03203-8

title: Feminizing adrenocortical adenoma in a girl from a resource-limited setting: a case report.

authors

- Elnaw EAA
- Ibrahim AAB

- Abdullah MA

journal: Journal of medical case reports

journalAbbr: J Med Case Rep

year: 2021

pubdate: 2021 Dec 21

volume: 15

issue: 1

pages: 605

abstract: BACKGROUND: An adrenocortical tumor is a rare tumor in pediatrics, which can be functional or nonfunctional. Functional tumors present with virilization, feminization, or hypercortisolism. Feminizing adrenal tumors, though rare in pediatrics, need to be excluded in any child presenting with features of feminization. CASE PRESENTATION: We report a case of a 4-year-old Sudanese girl who presented with gradually progressive bilateral breast enlargement and accelerated growth since the age of 6 months. The family had sought medical advice several times in numerous health facilities without much gain. Investigations showed pubertal luteinizing hormone levels, high estradiol E2, and dehydroepiandrosterone sulfate, with normal early morning cortisol level. Abdominal ultrasound revealed a right-sided hypoechoic suprarenal mass. Abdominal computed tomography scan showed a right adrenal mass. The diagnosis of feminizing adrenal neoplasm was confirmed and right adrenalectomy was done. Histopathological examination of the resected adrenal gland showed adrenocortical adenoma. The patient was started on gonadotrophin-releasing hormone agonist for secondary central precocious puberty. CONCLUSION: Adrenocortical tumors, though rare in pediatrics, are a documented cause of precocious puberty; biochemical and imaging screening protocol should be adopted for patients with precocious puberty, even in a resource-limited setting, for early detection and treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8690961/

Record 759

pmcid: PMC8056255

pmid: 33831630

doi: 10.1016/j.breast.2021.03.007

title: Frequency and outcomes of benign breast biopsies in trans women: A nationwide cohort study.

authors

- de Blok CJ
- Dijkman BA
- Wiepjes CM
- Konings IR
- Dreijerink KM
- Barbé E
- den Heijer M

journal: Breast (Edinburgh, Scotland)

journalAbbr: Breast

year: 2021

pubdate: 2021 Jun

volume: 57

issue:

pages: 118-122

abstract: No literature is available on the benign versus malignant breast lesion ratio in trans women (male sex assigned at birth, female gender identity). As hormone treatment in trans women results in breast tissue histologically comparable with cis (non-trans) women, breast pathology may be expected. Previously, an increased breast cancer risk compared with cis men have been observed. We aimed to investigate the frequency and outcomes of breast biopsies in trans women. Therefore, we retrospectively examined the medical files of 2616 trans women. To gain data on breast lesions, we linked our cohort to a national pathology database. In this study we found that 126 people (5%) had one or more breast biopsies (n = 139). Of these, 21 trans women had a breast biopsy before the start of hormone treatment, and 53 after the start of hormone treatment. Breast biopsies were performed predominantly because of abnormalities during physical examination (37%, n = 51/139 biopsies), or because of capsular formation or contraction (28%, n = 16/57 biopsies) in trans women with breast implants. The most common breast lesions after the start of hormone treatment were fibroadenomas (n = 20), breast cancer (n = 6), fibrosis (n = 5), cysts (n = 4), and infections (n = 4). The benign versus malignant breast biopsy ratio was 88:12, which is comparable to the ratio in cis women (90:10). This study shows breast lesions in a limited number of trans women. Since the indications and outcomes of biopsies in trans women were similar to those in cis women, it seems reasonable to follow breast care guidelines as developed for cis women.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8056255/

Record 760

pmcid: PMC8363464

pmid: 34395010

doi: 10.1155/2021/5570267

title: Functional Adrenocortical Adenoma in a Child with Beckwith-Wiedemann Syndrome.

authors

- Doya LJ
- Hassan NT
- Mansour HA
- Alkhalil MAA
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- Mansour N
- Abdallah A

journal: Case reports in pediatrics

journalAbbr: Case Rep Pediatr

year: 2021

pubdate: 2021

volume: 2021

issue:

pages: 5570267

abstract: Beckwith-Wiedemann syndrome (BWS) is a rare congenital condition characterized by complex overgrowth of different body parts. Children with Beckwith-Wiedemann syndrome, particularly those with hemihypertrophy, experience an increased risk of developing benign and malignant tumors. This case report presents an 18-month-old girl with features suggestive of Beckwith-Wiedemann syndrome who developed pubic hair, high levels of testosterone, and DHEAS with normal cortisol and progesterone levels. Computed tomography revealed a left adrenal mass. Histopathological examination of the resected mass showed an adrenocortical tumor. Her postoperative evaluation showed normal testosterone and DHEAS levels. Early diagnosis and detection of intra-abdominal neoplasms in infants with Beckwith-Wiedemann syndrome are essential to avoid serious clinical complications.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8363464/

Record 761

pmcid: PMC8053409

pmid: 33897993

doi: 10.1093/jscr/rjab050

title: Gestational gigantomastia-a rare entity complicated by life-threatening haemorrhage.

authors

- Neblett C
- Venugopal R
- Johnson M
- Mitchell D
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journal: Journal of surgical case reports

journalAbbr: J Surg Case Rep

year: 2021

pubdate: 2021 Apr

volume: 2021

issue: 4

pages: rjab050

abstract: Gestational gigantomastia is a psychologically and physically debilitating disease of unknown aetiology. Underlying diseases that present as gigantomastia should be excluded by a thorough workup. Most cases respond to the preferred approach: conservative management, as foetal viability and well-being is of significant importance. However, in those cases where the maternal mortality is at risk, the surgical approach is preferred. Life-threatening haemorrhage may occur and early recognition and treatment is paramount to outcome. A case of gestational gigantomastia complicated by life-threatening haemorrhage is presented and discussed.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8053409/

Record 762

pmcid: PMC8087397

pmid: 33968827

doi: 10.4322/acr.2021.252

title: Giant lactating adenoma.

authors

- Moulaz IR
- de Oliveira FSS
- da Silva EC
- Machado JM
- Santos MCLFS

journal: Autopsy & case reports

journalAbbr: Autops Case Rep

year: 2021

pubdate: 2021 Apr 15

volume: 11

issue:

pages: e2021252

abstract: Lactating adenoma is a rare benign breast lesion that most often presents as a small (up to 3 cm), solid, well-circumscribed, solitary, painless, mobile, lobulated mass. The highest incidence occurs in primiparous women (20 to 40 years old) during the third trimester of pregnancy. However, in the rare case presented herein, in addition to its giant size (more than 10 centimeters on palpation), this lactating adenoma is distinctive due to the presence of multiple nodules, poorly defined ultrasonographic margins, worrisome radiologic features, growth since early pregnancy, presence of infarction and association with chronic mastitis. From the clinical-radiologic perspective, the differential diagnoses included abscess associated with puerperal mastitis, phyllodes tumor, and galactocele. Biopsy was performed, and pathologic examination revealed the classic characteristics of lactating adenoma with multiple infarcted areas, leading to an unexpected confirmed case of giant lactating adenoma.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8087397/

Record 763

pmcid: PMC8488528

pmid: 34616560

doi: 10.1177/20551169211045640

title: Gynaecomastia in a male neutered cat with an adrenal tumour and associated hyperprogesteronism, hypercortisolism and hyperaldosteronism.

authors

- Yu J
- Lenord J
- Lau M
- Brunel L
- Gray R
- Donahoe SL
- Boland L

journal: JFMS open reports

journalAbbr: JFMS Open Rep

year: 2021

pubdate: 2021 Jul-Dec

volume: 7

issue: 2

pages: 20551169211045640

abstract: CASE SUMMARY: A 7-year-old male neutered domestic longhair cat was presented with chronic progressive gynaecomastia, polydipsia, polyphagia, weight loss and poor fur regrowth. Sexualised behavioural changes were not reported and virilisation was not present on physical examination. Pertinent haematology, biochemistry and urinalysis findings at the time of referral included mild hypokalaemia. Left adrenomegaly and mild prostatomegaly were identified on a CT scan. Evaluation of adrenal hormones with a low-dose dexamethasone suppression test, serum progesterone, testosterone, oestradiol, plasma aldosterone, renin, plasma metanephrine and normetanephrine measurement supported a diagnosis of hyperprogesteronism, hyperaldosteronism and hypercortisolism. Adrenalectomy was performed and histopathology was consistent

with an adrenocortical tumour. Clinical signs and hormone elevations resolved postoperatively. RELEVANCE AND NOVEL INFORMATION: To our knowledge, this is the second report of gynaecomastia secondary to an adrenal tumour in a male neutered cat and the first associated with hyperprogesteronism.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8488528/

Record 764

pmcid: PMC8007267

pmid: 33771805

doi: 10.3399/bjgp21X715577

title: Gynaecomastia: when and why to refer to specialist care.

authors

- Bromley HL
- Dave R
- Lord N
- Wright P
- Rowland M
- Gandhi A

journal: The British journal of general practice : the journal of the Royal College of General Practitioners

journalAbbr: Br J Gen Pract

year: 2021

pubdate: 2021 Apr

volume: 71

issue: 705

pages: 185-188

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8007267/

Record 765

pmcid: PMC7961508

pmid: 33806632

doi: 10.3390/jcm10051071

title: Gynecological Problems in Newborns and Infants.

authors

- Wróblewska-Seniuk K
- Jarząbek-Bielecka G
- Kędzia W

journal: Journal of clinical medicine

journalAbbr: J Clin Med

year: 2021

pubdate: 2021 Mar 4

volume: 10

issue: 5

pages:

abstract: Pediatric-adolescent or developmental gynecology has been separated from general gynecology because of the unique issues that affect the development and anatomy of growing girls and young women. It deals with patients from the neonatal period until maturity. There are not many gynecological problems that can be diagnosed in newborns; however, some are typical of the neonatal period. This paper aims to discuss the most frequent gynecological issues in the neonatal period.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7961508/

Record 766

pmcid: PMC7894277

pmid: 33643766

doi: 10.3205/iprs000151

title: Gynecomastia: a study to assess how students perceive this disease.

authors

- Al Jabr FA
- Zakaria OM
- Al Mulhim MA
- Alsuwailim AM
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journal: GMS Interdisciplinary plastic and reconstructive surgery DGPW

journalAbbr: GMS Interdiscip Plast Reconstr Surg DGPW

year: 2021

pubdate: 2021

volume: 10

issue:

pages: Doc01

abstract: Background : Gynecomastia is a benign proliferation of the glandular male breast tissue. Gynecomastia etiology might be physiological or non-physiological such as medications, chronic diseases (e.g. hypogonadism), or steroid supplements. Aim : The purpose of this study was to assess the knowledge and understanding of gynecomastia among medical students and which resources were used to gain their understanding regarding the disease. Methods : Data for this qualitative, questionnaire-based cross-sectional study was collected on the basis of our own study objectives and from available questionnaires with similar objectives. The questionnaire was composed of 26 questions divided into many items that were recorded including sociodemographic data, gynecomastia symptoms, and holistic perception of the problem by the students. Exclusion criteria included those who refused to participate in the study and did not complete the questionnaire. Statistical tests were taken significant at p-value ≤0.05. All analyses were performed using SPSS, version 21. Results : A total of 200 medical students participated in this study, among them more males than females (64% vs. 36%). We observed that medical students had significantly more moderate knowledge with teachers as their source of information on gynecomastia (p=0.028) while with books (p=0.005) and internet (p=0.041) as their sources of information they had significantly more a higher level of knowledge. Conclusions : Medical students have overall insufficient knowledge about gynecomastia especially in physical examination and treatment aspects. Therefore, gynecomastia is to be considered more thoroughly in the curriculum.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7894277/

Record 767

pmcid: PMC8564703

pmid: 33944894

doi: 10.1093/asj/sjab195

title: Hybrid Implant and Grafted Fat Breast Augmentation: Designing the Pathway to a Future With Breasts Free of Silicone Breast Implants.

authors

- Sforza M
- Spear S

journal: Aesthetic surgery journal

journalAbbr: Aesthet Surg J

year: 2021

pubdate: 2021 Oct 15

volume: 41

issue: 11

pages: NP1473-NP1485

abstract: BACKGROUND: Fat grafting in breast augmentation surgery is becoming increasingly popular, allowing surgeons to fill the gaps that implant augmentation alone cannot. However, one of the current issues surrounding fat grafting is the lack of standardization. OBJECTIVES: The objective of this study was to validate and summarize expert-based advice to help plastic surgeons better understand the benefits of utilizing fat grating to reduce implant size in their surgical planning. METHODS: This was a prospective study of fat grafting in 56 patients who underwent fat transfer together with silicone breast implants. A 3-dimensional planning system was used to plan the procedure and also to analyze the fat retention and resorption rates. Pictures served as a basis for a questionnaire in which both patients and physicians were asked to rate their satisfaction. RESULTS: Patients reported a satisfaction rate of "excellent" in 83.3% of cases and "good" in 16.7%. Physician satisfaction was rated as "excellent" in 84.5% of cases, "good" in 13.3%, and "fair" in 4.2% of cases. The mean fat volume grafted per breast was 113.63 mL. The mean absorption rate was 4.53%. CONCLUSIONS: The procedure presented consistent and reproducible results. Hybrid breast augmentation can help design a pathway for a future with breasts free of silicone. This information is particularly relevant for women who have their first breast augmentation at an early age and will probably have 2 to 5 additional surgeries in their lifetime due to the nature of current silicone breast technology.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8564703/

Record 768

pmcid: PMC8146565

pmid: 34092827

doi: 10.64719/pb.4396

title: Hyperprolactinemia, Clinical Considerations, and Infertility in Women on Antipsychotic Medications.

authors

- Edinoff AN
- Silverblatt NS
- Vervaeke HE
- Horton CC
- Girma E
- Kaye AD

- Kaye A
- Kaye JS
- Garcia AJ
- Neuchat EE
- Eubanks TN
- Varrassi G
- Viswanath O
- Urits I

journal: Psychopharmacology bulletin

journalAbbr: Psychopharmacol Bull

year: 2021

pubdate: 2021 Mar 16

volume: 51

issue: 2

pages: 131-148

abstract: Infertility, the inability to establish a clinical pregnancy after 12 months of regular unprotected sexual intercourse, is caused by a wide variety of both male and female factors. Infertility is estimated to affect between 8-12% of couples trying to conceive globally. Female factor infertility can be subdivided into the following broad categories: ovulatory dysfunction, fallopian tubal disease, uterine causes, and oocyte quality. Hyperprolactinemia causes ovulary dysfunction along with other hormonal abnormalities, such as decreased estrogen, which can lead to infertility. In this regard, antipsychotics are commonly used for both schizophrenia and bipolar disorder. The use of these medications can be associated with hyperprolactinemia and hyperprolactinemia associated infertility. Antipsychotic-induced hyperprolactinemia occurs through blockade of D 2 receptors on lactotroph cells of the anterior pituitary gland. Discontinuation of the hyperprolactinemia-inducing antipsychotic is an option, but this may worsen the patient's psychosis or mood. If antipsychotics are determined to be the culprit of infertility, the degree of hyperprolactinemia symptoms, length of treatment with the antipsychotic, and risk of relapse should be assessed prior to discontinuation, reduction, or switching of antipsychotic medications. The treatment of a women's mental health and her desire to have children should always be considered as treatment may influence fertility while on the medication.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8146565/

Record 769

pmcid: PMC8509256

pmid: 34640379

doi: 10.3390/jcm10194361

title: Hypogonadism in Adult Males with Prader-Willi Syndrome-Clinical Recommendations Based on a Dutch Cohort Study, Review of the Literature and an International Expert Panel Discussion.

authors

- Pellikaan K
- Ben Brahim Y
- Rosenberg AGW
- Davidse K
- Poitou C
- Coupaye M
- Goldstone AP

- Høybye C
- Markovic TP
- Grugni G
- Crinò A
- Caixàs A
- Eldar-Geva T
- Hirsch HJ
- Gross-Tsur V
- Butler MG
- Miller JL
- van den Berg SAA
- van der Lely AJ
- de Graaff LCG

journal: Journal of clinical medicine

journalAbbr: J Clin Med

year: 2021

pubdate: 2021 Sep 24

volume: 10

issue: 19

pages:

abstract: Prader-Willi syndrome (PWS) is a complex genetic syndrome characterized by hyperphagia, intellectual disability, hypotonia and hypothalamic dysfunction. Adults with PWS often have hormone deficiencies, hypogonadism being the most common. Untreated male hypogonadism can aggravate PWS-related health issues including muscle weakness, obesity, osteoporosis, and fatigue. Therefore, timely diagnosis and treatment of male hypogonadism is important. In this article, we share our experience with hypogonadism and its treatment in adult males with PWS and present a review of the literature. In order to report the prevalence and type of hypogonadism, treatment regimen and behavioral issues, we retrospectively collected data on medical interviews, physical examinations, biochemical measurements and testosterone replacement therapy (TRT) in 57 Dutch men with PWS. Fifty-six (98%) of the patients had either primary, central or combined hypogonadism. Untreated hypogonadism was associated with higher body mass index and lower hemoglobin concentrations. TRT was complicated by behavioral challenges in one third of the patients. Undertreatment was common and normal serum testosterone levels were achieved in only 30% of the patients. Based on the Dutch cohort data, review of the literature and an international expert panel discussion, we provide a practical algorithm for TRT in adult males with PWS in order to prevent undertreatment and related adverse health outcomes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8509256/

Record 770

pmcid: PMC8411320

pmid: 34189861

doi: 10.1002/npr2.12190

title: Hypoprolactinemia and hyperprolactinemia in male schizophrenia patients treated with aripiprazole and risperidone and their relationships with testosterone levels.

authors

- Tasaki M

- Yasui-Furukori N
- Yokoyama S
- Shinozaki M
- Sugawara N
- Shimoda K

journal: Neuropsychopharmacology reports

journalAbbr: Neuropsychopharmacol Rep

year: 2021

pubdate: 2021 Sep

volume: 41

issue: 3

pages: 379-384

abstract: AIM: Several reports have shown that risperidone increases prolactin concentrations, while aripiprazole decreases prolactin concentrations. The frequency of abnormal prolactin concentrations in patients with schizophrenia receiving these drugs is still unknown. Furthermore, although hyperprolactinemia leads to sexual dysfunction, the relationship between hyperprolactinemia and testosterone, which may be directly related to male sexual function, is not well understood. METHODS: The subjects were 94 male schizophrenia outpatients receiving risperidone or paliperidone (risperidone group) and 83 male schizophrenia outpatients receiving aripiprazole. We measured the serum prolactin and total and free testosterone concentrations. We compared the prolactin and testosterone levels in patients receiving risperidone or paliperidone and patients receiving aripiprazole. RESULTS: The average serum prolactin concentration was 27.5 ± 13.1 ng/mL for the risperidone group and 3.9 ± 3.5 ng/mL for the aripiprazole group, and the concentrations were significantly different ($P < .001$). Hypoprolactinemia was observed in 75% of the aripiprazole group and hyperprolactinemia in 65% of the risperidone group. A positive correlation between prolactin levels and the risperidone daily dose was found, whereas a negative correlation between prolactin levels and the aripiprazole daily dose was observed. In the risperidone group, total testosterone concentrations were correlated with age, while free testosterone concentrations were inversely correlated with age and prolactin levels. CONCLUSION: We found very common hyperprolactinemia and hypoprolactinemia in the risperidone or paliperidone group and aripiprazole group, respectively. Testosterone concentrations were associated with elevated prolactin levels in patients receiving risperidone or paliperidone. Further studies are needed to determine the clinical relevance of abnormal prolactin concentrations in male and female patients with schizophrenia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8411320/

Record 771

pmcid: PMC8803239

pmid: 35136627

doi: 10.1259/bjrcr.20210052

title: Idiopathic gigantomastia in a patient on polypharmacy.

authors

- Nguyen NJ
- Sanchez LM
- Yassa M
- David J
- El Khoury M

journal: BJR case reports

journalAbbr: BJR Case Rep

year: 2021

pubdate: 2021 Sep 8

volume: 7

issue: 5

pages: 20210052

abstract: Gigantomastia is an uncommon benign condition characterized by massive breast enlargement. It is most often due to hormonal imbalance secondary to puberty or pregnancy, or induced by a pharmacological agent but can also be idiopathic. Herein, we report a rare case of idiopathic gigantomastia in a 46-year-old female on antiepileptic multiple-drug therapy who underwent total bilateral mastectomy to relieve associated pain.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8803239/

Record 772

pmcid: PMC8448243

pmid: 34556919

doi: 10.1055/s-0041-1734358

title: Imaging Evaluation of Male Breast Masses with Histopathologic Correlation: A Case Series.

authors

- Vaidya TP
- Ramani SK

journal: The Indian journal of radiology & imaging

journalAbbr: Indian J Radiol Imaging

year: 2021

pubdate: 2021 Apr

volume: 31

issue: 2

pages: 360-365

abstract: The male breast can be afflicted with a wide spectrum of benign and malignant masses, similar to the female breast. A systematic radiological evaluation using mammography, ultrasonography, and when appropriate, magnetic resonance imaging, could aid this differentiation and provide clues to the diagnosis. In this article, we present six cases of male breast masses with an emphasis on the role of imaging in characterization and diagnosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8448243/

Record 773

pmcid: PMC8751073

pmid: 35010587

doi: 10.3390/ijerph19010328

title: Impact of Alcohol Consumption on Male Fertility Potential: A Narrative Review.

authors

- Finelli R
- Mottola F
- Agarwal A

journal: International journal of environmental research and public health

journalAbbr: Int J Environ Res Public Health

year: 2021

pubdate: 2021 Dec 29

volume: 19

issue: 1

pages:

abstract: Alcohol abuse disorder is a serious condition, implicating more than 15 million people aged 12 years and older in 2019 in the United States. Ethanol (or ethyl alcohol) is mainly oxidized in the liver, resulting in the synthesis of acetaldehyde and acetate, which are toxic and carcinogenic metabolites, as well as in the generation of a reductive cellular environment. Moreover, ethanol can interact with lipids, generating fatty acid ethyl esters and phosphatidylethanol, which interfere with physiological cellular pathways. This narrative review summarizes the impact of excessive alcohol consumption on male fertility by describing its metabolism and how ethanol consumption may induce cellular damage. Furthermore, the impact of alcohol consumption on hormonal regulation, semen quality, and genetic and epigenetic regulations is discussed based on evidence from animal and human studies, focusing on the consequences on the offspring. Finally, the limitations of the current evidence are discussed. Our review highlights the association between chronic alcohol consumption and poor semen quality, mainly due to the development of oxidative stress, as well as its genotoxic impact on hormonal regulation and DNA integrity, affecting the offspring's health. New landscapes of investigation are proposed for the identification of molecular markers for alcohol-associated infertility, with a focus on advanced OMICS-based approaches applied to the analysis of semen samples.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8751073/

Record 774

pmcid: PMC8446552

pmid: 34327874

doi: 10.1002/cam4.4170

title: Inflammatory breast cancer appearance at presentation is associated with overall survival.

authors

- Balema W
- Liu D
- Shen Y
- El-Zein R
- Debeb BG
- Kai M
- Overmoyer B
- Miller KD
- Le-Petross HT
- Ueno NT
- Woodward WA

journal: Cancer medicine

journalAbbr: Cancer Med

year: 2021

pubdate: 2021 Sep

volume: 10

issue: 18

pages: 6261-6272

abstract: BACKGROUND: Inflammatory breast cancer (IBC) is a clinical diagnosis. Here, we examined the association of a "classic" triad of clinical signs, swollen involved breast, nipple change, and diffuse skin change, with overall survival (OS). METHOD: Breast medical photographs from patients enrolled on a prospective IBC registry were scored by two independent reviewers as classic (triad above), not classic, and difficult to assign. Chi-squared test, Fisher's exact test, and Wilcoxon rank-sum test were used to assess differences between patient groups. Kaplan-Meier estimates and the log-rank test and Cox proportional hazard regression were used to assess the OS. RESULTS: We analyzed 245 IBC patients with median age 54 (range 26-81), M0 versus M1 status (157 and 88 patients, respectively). The classic triad was significantly associated with smoking, post-menopausal status, and metastatic disease at presentation ($p = 0.002$, 0.013, and 0.035, respectively). Ten-year actuarial OS for not classic and difficult to assign were not significantly different and were grouped for further analyses. Ten-year OS was 29.7% among patients with the classic sign triad versus 57.2% for non-classic ($p < 0.0001$). The multivariate Cox regression model adjusting for clinical staging ($p < 0.0001$) and TNBC status (<0.0001) demonstrated classic presentation score significantly associated with poorer OS time (HR 2.6, 95% CI 1.7-3.9, $p < 0.0001$). CONCLUSIONS: A triad of classic IBC signs independently predicted OS in patients diagnosed with IBC. Further work is warranted to understand the biology related to clinical signs and further extend the understanding of physical examination findings in IBC.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8446552/

Record 775

pmcid: PMC8012309

pmid: 32939601

doi: 10.1007/s00266-020-01949-y

title: Injection of Aquafilling(®) for Breast Augmentation Causes Inflammatory Responses Independent of Visible Symptoms.

authors

- Chalcarz M
- Żurawski J

journal: Aesthetic plastic surgery

journalAbbr: Aesthetic Plast Surg

year: 2021

pubdate: 2021 Apr

volume: 45

issue: 2

pages: 481-490

abstract: BACKGROUND: A major concern related to modern surgery is to evaluate and address the complications associated with breast enlargement using Aquafilling ® injection. This study aimed to assess the effect of Aquafilling ® injection on immune response in such patients. METHODS: For four patients who consulted a surgeon after receiving Aquafilling ® injection, medical history of the patients was taken; based on imaging examinations, Aquafilling ® was removed. Samples were processed for histopathological and immunohistochemical examination. For detecting tissue antigens in histopathological samples, monoclonal antibodies against CD3 (lymphocytes T), CD 20 (lymphocytes B), and CD68 (macrophages) were used. By analyzing the images, the number of immune cells (lymphocytes T, lymphocytes B, and macrophages) and immunohistochemical reaction area were semiquantitatively evaluated. RESULTS: Different clinical features were observed in each patient after receiving Aquafilling ® injection. In samples obtained from four patients, lymphocytes T (CD3), lymphocytes B (CD20), and macrophages (CD68) tissue expressions were observed.

Statistically significant variations in the number of lymphocytes B (CD20) and macrophages (CD68), and differentiation of immunohistochemical reaction area for lymphocytes T (CD3) and lymphocytes B (CD20) were observed. CONCLUSIONS: Inflammation is elevated in patients who received Aquafilling ® injection. Medical imaging should be carried out in all such patients even if there are no visible symptoms. Removal of Aquafilling ® can reduce the inflammation and risk of neoplastic progression in the patients. The influence of time elapsed since Aquafilling ® injection and intensity of immune response requires further validation. LEVEL OF EVIDENCE IV: This journal requires that authors assign a level of evidence to each article. For a full description of these Evidence-Based Medicine ratings, please refer to the Table of Contents or the online Instructions to Authors www.springer.com/00266 .

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8012309/

Record 776

pmcid: PMC7903291

pmid: 33609943

doi: 10.1016/j.ijscr.2021.01.075

title: Invasive pleomorphic lobular carcinoma of the breast with multiple metastases: A case report.

authors

- Chen X
- Lei L
- Tian C
- Ning P

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2021

pubdate: 2021 Mar

volume: 80

issue:

pages: 105581

abstract: INTRODUCTION AND IMPORTANCE: IPLC (Invasive Pleomorphic Lobular Carcinoma) accounts for less than 1% of breast cancer. To the best of our knowledge, this is one of the few reported cases in the IPLC with multiple metastases. The patient's general condition got improved after our treatment, which has a certain reference for the treatment of this kind of patient in the future. CASE PRESENTATION: A 42-year-old female with IPLC and multiple metastases. The IPLC malignant cells were positive for p120 (cytoplasmic) and human epidermal growth factor receptor 2, negative for estrogen receptor, progesterone receptor, and E-cadherin. There were nodular enhancement foci in the liver, which are considered the metastatic lesions of the breast, and the liver function was abnormal. Multiple metastatic lesions of the vertebral body, appendage of the whole spine, and sternum. And C7, T1, and T9 vertebrae showed compression fractures. CLINICAL DISCUSSION: IPLC has systemic metastasis which molecular typing by immunohistochemistry is HER-2 overexpression can choose chemotherapy combined with targeted therapy to prolong the survival time and improve the quality of life of patients. The patient was followed up. CONCLUSIONS: This paper reports a case of IPLC with multiple metastases and gives review literature. Our treatment of the patient can be a reference for other clinicians.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7903291/

Record 777

pmcid: PMC7910194

pmid: 33654621

doi: 10.7759/cureus.12944

title: It's Not Always Gynecomastia: A Case of Diffuse Large B-Cell Lymphoma Presenting as Breast Mass in a Male Patient.

authors

- Leon A
- Way AR
- Sharma S
- Letter H

journal: Cureus

journalAbbr: Cureus

year: 2021

pubdate: 2021 Jan 27

volume: 13

issue: 1

pages: e12944

abstract: We report a case of diffuse large B-cell breast lymphoma that presented as a palpable breast lump in a male patient evaluated with digital mammography and targeted breast ultrasound (US) but ultimately confirmed by US-guided core needle biopsy. We will discuss the appropriate workup algorithm for a palpable breast lump in a male patient as outlined by the American College of Radiology (ACR) Appropriateness Criteria. While gynecomastia is the most common reason for a new palpable breast lump in a male patient, male breast cancer (including lymphoma and metastasis) can have a similar appearance on imaging. Our goal is to provide clarity on proper radiographic assessment protocols and imaging features of palpable breast masses in men by emphasizing the role of anatomical location and symmetry in distinguishing it from more common causes, such as gynecomastia, in future diagnostic imaging scenarios.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7910194/

Record 778

pmcid: PMC8336914

pmid: 34354904

doi: 10.1007/s40674-020-00168-5

title: Juvenile Dermatomyositis: New Clues to Diagnosis and Therapy.

authors

- Pachman LM
- Nolan BE
- DeRanieri D
- Khojah AM

journal: Current treatment options in rheumatology

journalAbbr: Curr Treatm Opt Rheumatol

year: 2021

pubdate: 2021 Mar

volume: 7

issue: 1

pages: 39-62

abstract: PURPOSE OF REVIEW: To identify clues to disease activity and discuss therapy options. RECENT

FINDINGS: The diagnostic evaluation includes documenting symmetrical proximal muscle damage by exam and MRI, as well as elevated muscle enzymes-aldolase, creatine phosphokinase, LDH, and SGOT-which often normalize with a longer duration of untreated disease. Ultrasound identifies persistent, occult muscle inflammation. The myositis-specific antibodies (MSA) and myositis-associated antibodies (MAA) are associated with specific disease course variations. Anti-NXP-2 is found in younger children and is associated with calcinosis; anti-TIF-1γ+ juvenile dermatomyositis has a longer disease course. The diagnostic rash-involving the eyelids, hands, knees, face, and upper chest-is the most persistent symptom and is associated with microvascular compromise, reflected by loss of nailfold (periungual) end row capillaries. This loss is associated with decreased bioavailability of oral prednisone; the bioavailability of other orally administered medications should also be considered. At diagnosis, at least 3 days of intravenous methyl prednisolone may help control the HLA-restricted and type 1/2 interferon-driven inflammatory process. The requirement for avoidance of ultraviolet light exposure mandates vitamin D supplementation. SUMMARY: This often chronic illness targets the cardiovascular system; mortality has decreased from 30 to 1-2% with corticosteroids. New serological biomarkers indicate occult inflammation: ↑CXCL-10 predicts a longer disease course. Some biologic therapies appear promising.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8336914/

Record 779

pmcid: PMC8278142

pmid: 34285730

doi: 10.1016/j.radcr.2021.06.006

title: Juvenile granulosa cell tumor diagnosed in 6-month-old infant with precocious puberty.

authors

- Hansen R
- Lewis A
- Sullivan C
- Hirsig L

journal: Radiology case reports

journalAbbr: Radiol Case Rep

year: 2021

pubdate: 2021 Sep

volume: 16

issue: 9

pages: 2609-2613

abstract: Juvenile granulosa cell tumor is a rare tumor diagnosed in children, which can present with precocious puberty. We have reported a case of a 6-month-old female patient who presented with precocious puberty. Abdominopelvic imaging revealed a large mixed cystic and solid mass, with internal solid enhancement, and restricted diffusion. At surgery, mass was confirmed to arise from the left ovary, and histopathology confirmed juvenile granulosa cell tumor. We provide a literature review of juvenile granulosa cell tumor and discuss imaging characteristics of this diagnosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8278142/

Record 780

pmcid: PMC8104375

pmid: 33961665

doi: 10.1371/journal.pone.0251302

title: Knowledge and beliefs about the use/abuse of oral contraceptive pills among males: A mixed-method explanatory sequential study in community pharmacy settings.

authors

- Barakat M
- Al-Qudah R
- Akour A
- Abu-Asal M
- Thiab S
- Dallal Bashi YH

journal: PloS one

journalAbbr: PLoS One

year: 2021

pubdate: 2021

volume: 16

issue: 5

pages: e0251302

abstract: BACKGROUND: Oral contraceptive pills (OCPs) are considered one of the most important birth control methods globally. However, these pills were designed for female administration rather than males. This study was designed to investigate patterns of OCPs use and abuse among Jordanian males, according to the community pharmacists' observations. METHOD: A mixed-method explanatory sequential study was conducted using an online self-administered survey, followed by semi-structured in-depth interviews for registered pharmacists, assistant pharmacists and pharmacy interns. The interviews were utilized using a conceptual framework. Inductive thematic analysis and descriptive/regression analyses were completed using Nvivo and SPSS, respectively. RESULTS: A total of 158 questionnaire responses and 22 interviews were included in our analysis. Around half (48.4%) of the questionnaire responses confirmed that males could use OCPs for hair growth enhancement, muscle gain and acne treatment 12.7%, 31.7% and 4.4%, respectively. Through the interviews, the participating pharmacists highlighted that males use OCPs mostly for bodybuilding purposes, according to recommendations by their coaches at the gym. The most abused OCPs containing estrogen (Ethinyl estradiol) and progestins (Drospirenone or Levonorgestrel). CONCLUSION: This study provided insight into unexpected uses of OCPs by males in Jordan. Community pharmacists have a crucial role in the management of OCPs use and abuse. However, restricted regulations and monitoring must be released and implemented on the community to limit such practices.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8104375/

Record 781

pmcid: PMC8258880

pmid: 34268093

doi: 10.21037/gs-20-853

title: Large cell anaplastic lymphoma associated with breast implant: a rare case report presentation and discussion of possible management.

authors

- Tripodi D
- Amabile MI
- Varanese M
- D'Andrea V

- Sorrenti S
- Cannistrà C

journal: Gland surgery

journalAbbr: Gland Surg

year: 2021

pubdate: 2021 Jun

volume: 10

issue: 6

pages: 2076-2080

abstract: Anaplastic large cell lymphomas (ALCL) are a rare type of primary breast lymphoma. The association between breast implants and ALCL was first described in 1997. Breast implant associated (BIA)-ALCL arises from the inflammatory T cells surrounding the fibrous capsule, and most tumors are in situ . Here we present the case of a 60-year-old woman with ALCL following bilateral silicone breast prosthesis implantation for aesthetic reason. The patient presented at our observation 7 years following the first surgery reporting a sport trauma in the right thoracic region with breast enlargement and tenderness, complaining breast pain at the palpation of the right breast. Imaging study showed a right fluid collection surrounding the affected breast implant. For this reason, the patient underwent bilateral complete capsulectomy (surgical specimen histologically analyzed and resulted negative for ALCL) and implantation of new breast silicone prosthesis. In 10 months, a progressive relapse of the symptoms with a right peri-implant fluid collection restauration was documented and bilateral surgical removal of breast prostheses with right peri-implant capsular biopsy were performed. The histological analysis of the peri-implant capsule laid for ALCL cellules and a new radical surgical excision of bilateral periprothesic capsule was performed followed by adjuvant radiotherapy for a skin relapse of the disease with a subsequent complete remission. No further relapse was observed afterwards. This clinical case raised a series of diagnostic and therapeutic issues highlighting similarities with previous BIA-ALCL reports, but also some peculiarities that deserve to be further investigated and described in order to make an early recognition of the disease for the proper management and treatment of the patient. Due to the rarity of these tumors, rapidly diagnosing a BIA-ALCL case, not neglecting other associated symptoms or signs, is essential to ensure timely and safe treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8258880/

Record 782

pmcid: PMC8632385

pmid: 34858564

doi: 10.1155/2021/7956184

title: Machine Learning-Based Gynecologic Tumor Diagnosis and Its Postoperative Incisional Infection Influence Factor Analysis.

authors

- Shen Q
- Wang L

journal: Journal of healthcare engineering

journalAbbr: J Healthc Eng

year: 2021

pubdate: 2021

volume: 2021

issue:

pages: 7956184

abstract: Various factors influencing postoperative incisional infection in gynecologic tumors were analyzed, and the value of quality nursing intervention was studied. In this study, 74 surgically treated gynecologic tumor patients were randomly selected from within the hospital as the study population and were divided into study and control groups. For this purpose, the whole-group random sampling method is utilized to compare the postoperative incisional infection rates of the two groups, analyze their influencing factors, and develop quality nursing interventions. In this paper, a breast cancer diagnosis prediction model was developed by combining the self-attentive mechanism. The preprocessing work such as data quantification and normalization was performed first which is followed by adding the preprocessed data to the self-attentive mechanism. This model has solved the problem that recurrent neural networks (RNNs) could not extract and calculate the features at the same time. Likewise, it has solved the drawback that the RNN could not consider global features at the same time when extracting the features, and then, the feature matrix extracted by the self-attentive mechanism was added to the adaptive neural network. The adaptive neural network model for breast cancer diagnosis prediction was constructed and, finally, relevant parameters of the adaptive neural network model were adjusted according to different tasks to make the model performance optimal. Experimental results showed that the postoperative incision infection rate of patients in the study group was 2.70%, which was significantly lower than that of 21.62% in the control group ($P < 0.05$). Likewise, operation time, operation method, hospitalization time, preoperative fever, diabetes mellitus, and anemia were the main influencing factors of postoperative incision infection in women with gynecologic tumors. The time of surgery, surgical method, long hospital stay, preoperative fever, diabetes, and anemia are the main factors that lead to postoperative incisional infection in female gynecologic tumor patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8632385/

Record 783

pmcid: PMC8254943

pmid: 34217263

doi: 10.1186/s12902-021-00803-9

title: Madelung's disease in a non-alcoholic Ethiopian male patient mistaken for obesity: a case report.

authors

- Zewde YZ

journal: BMC endocrine disorders

journalAbbr: BMC Endocr Disord

year: 2021

pubdate: 2021 Jul 3

volume: 21

issue: 1

pages: 142

abstract: BACKGROUND: Madelung's disease (MD) is a rare disorder of fat storage characterized by the presence of diffuse, symmetrical deposition of subcutaneous fat around the neck, shoulder, arm, trunk and thigh. Although its cause is not fully understood, this benign condition is commonly presented among adult males with Mediterranean origin and history of alcohol abuse. Patients often presents with compression of vital structures, cosmetic disfigurement and associated psychosocial problems and systemic comorbidities. It is often under-recognized by physicians, possibly due to obliviousness of the condition and often misdiagnosed as obesity. CASE PRESENTATION: We present a 65-year-old non-alcoholic black Ethiopian man, presented with a slowly growing body fat in his trunk and proximal limbs associated by multiple joint and back pain which got worse recently. He denied any history of chronic alcohol use. On examination, huge, bilateral, non-tender, soft, globular masses in his torso, shoulder, arm and thigh with bilateral breast enlargement. On investigation his biochemical profile was normal except hyperuricemia (10.6 mg/dl). Imaging of the cervical and lumbar vertebrae showed excess subcutaneous fat depositions with degenerative disc disease. Biopsy from the mass revealed

non-encapsulated lipoma and he was diagnosed with type II MD. We treated his pain with supportive therapy and discharged in stable condition. The patient deferred surgical treatment. CONCLUSIONS: Madelung's disease is often reported among white adult males with chronic alcoholism. However, our case reported a black man without the typical risk factor which was misdiagnosed as obesity. Hence, clinicians should be aware of MD and need to consider it in their differential diagnosis when encountered with a patient having progressive centripetal fat deposition with or without a history of alcoholism and systemic comorbidities. As early detection of this disorder helps to avoid diagnostic delays and prevent complications through timely interventions which will in turn improves patient quality of life.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8254943/

Record 784

pmcid: PMC8462200

pmid: 34621881

doi: 10.12998/wjcc.v9.i27.8199

title: Madelung's disease with alcoholic liver disease and acute kidney injury: A case report.

authors

- Wu L
- Jiang T
- Zhang Y
- Tang AQ
- Wu LH
- Liu Y
- Li MQ
- Zhao LB

journal: World journal of clinical cases

journalAbbr: World J Clin Cases

year: 2021

pubdate: 2021 Sep 26

volume: 9

issue: 27

pages: 8199-8206

abstract: BACKGROUND: Madelung's disease (MD) is a rare disorder of lipid metabolism, characterized by the growth of unencapsulated masses of adipose tissue symmetrically deposited around the neck, shoulders, or other sites around the body. Its pathological mechanism is not yet known. One of the most common comorbidities in MD patients is liver disease, especially chronic alcoholic liver disease (CALD); however, no reports exist of acute kidney injury (AKI) with MD. CASE SUMMARY: We report a 60-year-old man who presented with complaint of edema in the lower limbs that had persisted for 3 d. Physical examination showed subcutaneous masses around the neck, and history-taking revealed the masses to have been present for 2 years and long-term heavy drinking. Considering the clinical symptoms, along with various laboratory test results and imaging characteristics, a diagnosis was made of MD with acute exacerbation of CALD and AKI. The patient was treated with liver function protection and traditional Chinese medicine, without surgical intervention. He was advised to quit drinking. After 10 d, the edema had subsided, renal function indicators returned to normal, liver function significantly improved, and size of subcutaneous masses remained stable. CONCLUSION: In MD, concomitant liver or kidney complications are possible and monitoring of liver and kidney functions can be beneficial.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8462200/

Record 785

pmcid: PMC8010557

pmid: 33544650

doi: 10.1259/bjr.20201013

title: Magnetic resonance imaging in the evaluation of pathologic nipple discharge: indications and imaging findings.

authors

- Samreen N
- Madsen LB
- Chacko C
- Heller SL

journal: The British journal of radiology

journalAbbr: Br J Radiol

year: 2021

pubdate: 2021 Apr 1

volume: 94

issue: 1120

pages: 20201013

abstract: Pathologic nipple discharge (PND) is typically unilateral, spontaneous, involves a single duct, and is serous or bloody in appearance. In patients with PND, breast MRI can be helpful as an additional diagnostic tool when conventional imaging with mammogram and ultrasound are negative. MRI is able to detect the etiology of nipple discharge in 56-61% of cases when initial imaging with mammogram and ultrasound are negative. Advantages to using MRI in evaluation of PND include good visualization of the retroareolar breast and better evaluation of posterior lesions which may not be well evaluated on mammograms and galactograms. It is also less invasive compared to central duct excision. Papillomas and nipple adenomas are benign breast masses that can cause PND and are well visualized on MRI. Ductal ectasia, and infectious etiologies such as mastitis, abscess, and fistulas are additional benign causes of PND that are well evaluated with MRI. MRI is also excellent for evaluation of malignant causes of PND including Paget's disease, ductal carcinoma in-situ and invasive carcinoma. MRI's high negative predictive value of 87-98.2% is helpful in excluding malignant etiologies of PND.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8010557/

Record 786

pmcid: PMC8036380

pmid: 33800580

doi: 10.3390/ijerph18073502

title: Medical Knowledge, Religious Beliefs, and Free Will: Attitudes and Opinions of Various Undergraduate Female Respondents Regarding Oral Contraception. A Questionnaire-Based Study.

authors

- Ősz BE
- Ștefănescu R
- Tero-Vescan A
- Sălcudean A
- Boca CD

- Jîtcă G
- Vari CE

journal: International journal of environmental research and public health

journalAbbr: Int J Environ Res Public Health

year: 2021

pubdate: 2021 Mar 28

volume: 18

issue: 7

pages:

abstract: The decision to use oral contraception varies and is based on several considerations: Personal reasons, the evaluation of the benefit/ risk ratio, and religious beliefs. In this research, a questionnaire was distributed to 422 female students from the George Emil Palade University of Medicine, Pharmacy, Science, and Technology of Târgu Mureş, Romania (UMPhST Târgu Mureș, Romania), aged between 19 and 24 years old. The first endpoint of the study was to evaluate the use of hormonal contraception by the sexually active female population. The second endpoint was to assess the degree of awareness of the benefit/risk ratio of oral contraceptive use. The third endpoint was to evaluate the influence of religious beliefs regarding the decision to use this type of pharmaceutical product. Our results show that only a small percentage of students chose to use oral contraceptive pills (OCP). Fortunately, most of the respondents were well-informed and used a particular contraceptive drug based on a healthcare professional's recommendation. Another aspect that emphasizes the choice of contraceptive method is the religious affiliation, which could influence the decision to use OCP. For the students with medical knowledge, the advice of a healthcare professional seems to be quite important, because they are aware of the risks of improper use of OCP. Although religious doctrines affect the decision to use oral contraception, this is not always taken into account, as the use of OCP is a personal decision.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8036380/

Record 787

pmcid: PMC7919692

pmid: 33672087

doi: 10.3390/molecules26041032

title: Medicinal Use of Testosterone and Related Steroids Revisited.

authors

- Tauchen J
- Jurášek M
- Huml L
- Rimpelová S

journal: Molecules (Basel, Switzerland)

journalAbbr: Molecules

year: 2021

pubdate: 2021 Feb 15

volume: 26

issue: 4

pages:

abstract: Testosterone derivatives and related compounds (such as anabolic-androgenic steroids-AAS) are frequently misused by athletes (both professional and amateur) wishing to promote muscle development and

strength or to cover AAS misuse. Even though these agents are vastly regarded as abusive material, they have important pharmacological activities that cannot be easily replaced by other drugs and have therapeutic potential in a range of conditions (e.g., wasting syndromes, severe burns, muscle and bone injuries, anemia, hereditary angioedema). Testosterone and related steroids have been in some countries treated as controlled substances, which may affect the availability of these agents for patients who need them for therapeutic reasons in a given country. Although these agents are currently regarded as rather older generation drugs and their use may lead to serious side-effects, they still have medicinal value as androgenic, anabolic, and even anti-androgenic agents. This review summarizes and revisits the medicinal use of compounds based on the structure and biological activity of testosterone, with examples of specific compounds. Additionally, some of the newer androgenic-anabolic compounds are discussed such as selective androgen receptor modulators, the efficacy/adverse-effect profiles of which have not been sufficiently established and which may pose a greater risk than conventional androgenic-anabolic agents.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7919692/

Record 788

pmcid: PMC7870600

pmid: 33394204

doi: 10.1007/s10552-020-01380-3

title: Men's experiences of radiotherapy treatment for localized prostate cancer and its long-term treatment side effects: a longitudinal qualitative study.

authors

- Sutton E
- Lane JA
- Davis M
- Walsh EI
- Neal DE
- Hamdy FC
- Mason M
- Staffurth J
- Martin RM
- Metcalfe C
- Peters TJ
- Donovan JL
- Wade J

journal: Cancer causes & control : CCC

journalAbbr: Cancer Causes Control

year: 2021

pubdate: 2021 Mar

volume: 32

issue: 3

pages: 261-269

abstract: PURPOSE: To investigate men's experiences of receiving external-beam radiotherapy (EBRT) with neoadjuvant Androgen Deprivation Therapy (ADT) for localized prostate cancer (LPCa) in the ProtecT trial. METHODS: A longitudinal qualitative interview study was embedded in the ProtecT RCT. Sixteen men with clinically LPCa who underwent EBRT in ProtecT were purposively sampled to include a range of socio-

demographic and clinical characteristics. They participated in serial in-depth qualitative interviews for up to 8 years post-treatment, exploring experiences of treatment and its side effects over time. RESULTS: Men experienced bowel, sexual, and urinary side effects, mostly in the short term but some persisted and were bothersome. Most men downplayed the impacts, voicing expectations of age-related decline, and normalizing these changes. There was some reticence to seek help, with men prioritizing their relationships and overall health and well-being over returning to pretreatment levels of function. Some unmet needs with regard to information about treatment schedules and side effects were reported, particularly among men with continuing functional symptoms. CONCLUSIONS: These findings reinforce the importance of providing universal clear, concise, and timely information and supportive resources in the short term, and more targeted and detailed information and care in the longer term to maintain and improve treatment experiences for men undergoing EBRT.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7870600/

Record 789

pmcid: PMC8058137

pmid: 33877461

doi: 10.1186/s13244-021-00991-x

title: Mimickers of breast malignancy: imaging findings, pathologic concordance and clinical management.

authors

- Guirguis MS
- Adrada B
- Santiago L
- Candelaria R
- Arribas E

journal: Insights into imaging

journalAbbr: Insights Imaging

year: 2021

pubdate: 2021 Apr 20

volume: 12

issue: 1

pages: 53

abstract: Many benign breast entities have a clinical and imaging presentation that can mimic breast cancer. The purpose of this review is to illustrate the wide spectrum of imaging features that can be associated with benign breast diseases with an emphasis on the suspicious imaging findings associated with these benign conditions that can mimic cancer. As radiologic-pathologic correlation can be particularly challenging in these cases, the radiologist's familiarity with these benign entities and their imaging features is essential to ensure that a benign pathology result is accepted as concordant when appropriate and that a suitable management plan is formulated.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8058137/

Record 790

pmcid: PMC8696644

pmid: 35047120

doi: 10.4239/wjd.v12.i12.2058

title: Molecular diagnosis of Kallmann syndrome with diabetes by whole exome sequencing and bioinformatic approaches.

authors

- Sun SS
- Wang RX

journal: World journal of diabetes

journalAbbr: World J Diabetes

year: 2021

pubdate: 2021 Dec 15

volume: 12

issue: 12

pages: 2058-2072

abstract: BACKGROUND: Kallmann syndrome (KS) is a hypogonadotropic hypogonadism accompanied by anosmia or hyposmia. It is associated with the low secretion of gonadotropins which can lead to other abnormal endocrine metabolism disorders such as diabetes. Through genetic and molecular biological methods, more than 10 KS pathogenic genes have been found. AIM: To identify the existing mutation sites of KS with diabetes and reveal the relationship between genotype and phenotype. METHODS: We studied KS pathogenesis through high-throughput exome sequencing on four diabetes' patients with KS for screening the potential pathogenic sites and exploring the genotype-phenotype correlation. Clinical data and peripheral blood samples were collected from the patients. White blood cells were separated and genomic DNA was extracted. High-throughput sequencing of all exons in the candidate pathogenic genes of probands was performed, and the results obtained were analyzed. RESULTS: Sequencing revealed mutations in the KLB p.T313M, ANOS1 p.C172F, and IGSF10 gene (p.Lys1819Arg and p.Arg1035Thr) at different sites, which may have been associated with disease onset. CONCLUSION: The diagnosis of KS is challenging, especially in early puberty, and the clinical manifestations reflect physical delays in development and puberty. Timely diagnosis and treatment can induce puberty, thereby improving sexual, bone, metabolic and mental health.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8696644/

Record 791

pmcid: PMC9149696

pmid: 33896789

doi: 10.15537/smj.2021.42.5.20200459

title: Peri-areolar double-pedicle technique in the treatment of iatrogenic gynecomastia.

authors

- Cannistrà C
- Al-Shaqsi Y

journal: Saudi medical journal

journalAbbr: Saudi Med J

year: 2021

pubdate: 2021 May

volume: 42

issue: 5

pages: 574-577

abstract: OBJECTIVES: To describe the surgical approach to the treatment of iatrogenic gynecomastia via peri-areolar incision. METHODS: In this study, we describe our experience in the surgical management of iatrogenic hormonal gynecomastia with our technique described for idiopathic gynecomastia. We treated 70 patients with gynecomastia at a plastic surgery unit between 2000 and 2016. Ten of these patients had a painful hypertrophic

mammary gland with grade III gynecomastia secondary to hormone therapy after prostate cancer. RESULTS: The presence of a very discreet scar around the areola often has very interesting aesthetic results. Areolar tissue sensation was completely preserved in 40% of patients. No short-term complications were observed; in follow-up, only one case of keloid scars was noted on a patient with dark skin, and only one case of areolar necrosis on a patient with critical breast ptosis. CONCLUSION: This technique is feasible. It restores almost normal anatomy structure with minimal scarring and preserves good regional neurovascular supply, with only 60% altered areolar sensation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9149696/

Record 792

pmcid: PMC8329252

pmid: 34354591

doi: 10.3389/fphar.2021.699044

title: Phytochemicals as Potential Chemopreventive and Chemotherapeutic Agents for Emerging Human Papillomavirus-Driven Head and Neck Cancer: Current Evidence and Future Prospects.

authors

- Aggarwal N
- Yadav J
- Chhakara S
- Janjua D
- Tripathi T
- Chaudhary A
- Chhokar A
- Thakur K
- Singh T
- Bharti AC

journal: Frontiers in pharmacology

journalAbbr: Front Pharmacol

year: 2021

pubdate: 2021

volume: 12

issue:

pages: 699044

abstract: Head and neck cancer (HNC) usually arises from squamous cells of the upper aerodigestive tract that line the mucosal surface in the head and neck region. In India, HNC is common in males, and it is the sixth most common cancer globally. Conventionally, HNC attributes to the use of alcohol or chewing tobacco. Over the past four decades, portions of human papillomavirus (HPV)-positive HNC are increasing at an alarming rate. Identification based on the etiological factors and molecular signatures demonstrates that these neoplastic lesions belong to a distinct category that differs in pathological characteristics and therapeutic response. Slow development in HNC therapeutics has resulted in a low 5-year survival rate in the last two decades. Interestingly, HPV-positive HNC has shown better outcomes following conservative treatments and immunotherapies. This raises demand to have a pre-therapy assessment of HPV status to decide the treatment strategy. Moreover, there is no HPV-specific treatment for HPV-positive HNC patients. Accumulating evidence suggests that phytochemicals are promising leads against HNC and show potential as adjuvants to chemoradiotherapy in HNC. However, only a few of these phytochemicals target HPV. The aim of the present article was to collate data on various leading phytochemicals that have shown promising results in the

prevention and treatment of HNC in general and HPV-driven HNC. The review explores the possibility of using these leads against HPV-positive tumors as some of the signaling pathways are common. The review also addresses various challenges in the field that prevent their use in clinical settings.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8329252/

Record 793

pmcid: PMC8538984

pmid: 34681876

doi: 10.3390/ijms222011218

title: Phytoestrogens (Resveratrol and Equol) for Estrogen-Deficient Skin-Controversies/Misinformation versus Anti-Aging In Vitro and Clinical Evidence via Nutraceutical-Cosmetics.

authors

- Lephart ED

journal: International journal of molecular sciences

journalAbbr: Int J Mol Sci

year: 2021

pubdate: 2021 Oct 18

volume: 22

issue: 20

pages:

abstract: The overarching theme for this review is perspective. Superfoods (a marketing term for fruits and vegetables, etc.) have a positive connotation, while many superfoods contain phytoestrogens, a term that is alarming to the public and has a negative connotation because phytoestrogens are endocrine-disruptors, even though they are strong antioxidants that have many health benefits. To understand phytoestrogens, this paper provides a brief summary of the characteristics of: (a) estrogens, (b) estrogen receptors (ER), (c) estrogen-deficient skin, (d) how perspective(s) get off track, (e) phytoestrogen food sources, and (f) misconceptions of phytoestrogens and food safety, in general, that influence person(s) away from what is true. Finally, a brief history of cosmetics to nutraceuticals is covered plus the characteristics of phytoestrogens, resveratrol and equol on: (g) estrogen receptor binding, (h) topical and oral dosing, and (i) in vitro, molecular mechanisms and select clinical evidence, where both phytoestrogens (resveratrol and equol) demonstrate promising applications to improve skin health is presented along with future directions of nutraceuticals. Perspective is paramount in understanding the controversies associated with superfoods, phytoestrogens, and endocrine-disruptors because they have both positive and negative connotations. Everyone is exposed to and consumes these molecules everyday regardless of age, gender, or geographic location around the world, and how we understand this is a matter of perspective.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8538984/

Record 794

pmcid: PMC9431967

pmid: 36238888

doi: 10.3348/jksr.2021.0049

title: Postpartum Galactocele in Augmented Breast after Using Breast Pump Mimicking Breast Implant-Associated Anaplastic Large Cell Lymphoma: A Case Report.

authors

- Lee GY
- Shin GW

- Park YM
- Lee A
- Park HY
- Lee YJ
- Han JY

journal: Taehan Yongsang Uihakhoe chi

journalAbbr: Taehan Yongsang Uihakhoe Chi

year: 2021

pubdate: 2021 Nov

volume: 82

issue: 6

pages: 1570-1574

abstract: Post-partum galactocele is a common benign disease among breastfeeding women, whereas retromammary and peri-implant galactocele are relatively rare conditions. Herein, a 34-year-old, 1 month-postpartum female, who had augmentation mammoplasty and a 1-month history of breast pump use, presented with left breast enlargement for 2 weeks. An initial left breast US revealed hyperechoic peri-implant fluid collection. Additional US-guided fine needle aspiration was done using a 21G-needle, draining the milk component in the process, and cytologic results revealed numerous crystals, suggestive of galactocele. Various diseases, especially breast implant-associated anaplastic large cell lymphoma, can cause peri-implant fluid collection in an augmented breast. Thus, correlating imaging features with clinical information and cytologic analysis plays an important role in appropriate management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9431967/

Record 795

pmcid: PMC8141617

pmid: 34041247

doi: 10.3389/fcell.2021.672890

title: Premature Ovarian Insufficiency: Past, Present, and Future.

authors

- Chon SJ
- Umair Z
- Yoon MS

journal: Frontiers in cell and developmental biology

journalAbbr: Front Cell Dev Biol

year: 2021

pubdate: 2021

volume: 9

issue:

pages: 672890

abstract: Premature ovarian insufficiency (POI) is the loss of normal ovarian function before the age of 40 years, a condition that affects approximately 1% of women under 40 years old and 0.1% of women under 30 years old. It is biochemically characterized by amenorrhea with hypoestrogenic and hypergonadotropic conditions, in some cases, causing loss of fertility. Heterogeneity of POI is registered by genetic and non-genetic causes, such as autoimmunity, environmental toxins, and chemicals. The identification of possible causative genes and selection

of candidate genes for POI confirmation remain to be elucidated in cases of idiopathic POI. This review discusses the current understanding and future prospects of heterogeneous POI. We focus on the genetic basis of POI and the recent studies on non-coding RNA in POI pathogenesis as well as on animal models of POI pathogenesis, which help unravel POI mechanisms and potential targets. Despite the latest discoveries, the crosstalk among gene regulatory networks and the possible therapies targeting the same needs to explore in near future.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8141617/

Record 796

pmcid: PMC8265982

pmid:

doi: 10.1210/jendso/bvab048.1231

title: Prolactinoma in Two Brothers - Unidentified Genetic Cause Versus Co-Incidence?

authors

- Gundluru R
- Liu JC
- Parimi JTN
- Naha S
- Gusov T
- Syed H

journal: Journal of the Endocrine Society

journalAbbr: J Endocr Soc

year: 2021

pubdate: 2021 May 3

volume: 5

issue: Suppl 1

pages: A604

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8265982/

Record 797

pmcid: PMC8064852

pmid: 33927534

doi: 10.1055/s-0041-1726615

title: Pseudocyesis Leading to Folie-à-Deux.

authors

- Nagdive AB
- Bhainsora RS
- Fernandes R
- Behere PB
- Sethi S

journal: Journal of neurosciences in rural practice

journalAbbr: J Neurosci Rural Pract

year: 2021

pubdate: 2021 Apr

volume: 12

issue: 2

pages: 419-423

abstract: Pseudocyesis or false belief of pregnancy is the emergence of classical manifestations of pregnancy-nausea, breast enlargement and pigmentation, abdominal distention, amenorrhea, and labor pains-in a nonpregnant woman. It is a multifactorial disease and its development is influenced by many different elements such as neuroendocrine, social, psychodynamic, and cultural issues. "Folie-à-deux," is shared psychotic disorder, describes a syndrome in which delusion is transferred to another person who is more susceptible. Both individuals are closely related or know each other for a long time and typically live together in relative social isolation. In its commonest form, the individual who first develops the delusion (the primary case) is often chronically ill and typically is the dominant member in a close relationship with a more suggestible person (the secondary case) who also develops the delusion. Treatment options should also be kept in mind as antipsychotics themselves can increase prolactin levels and can lead to amenorrhoea and galactorrhea and can further strengthen patient's belief about her pregnancy. This case highlights that the most important therapeutic step in the treatment of folie-à-deux is separation of the inducer and the induced. Here we describe a case of folie-à-deux of a married couple in which the female had delusional pregnancy while the husband shared and supported her delusion against substantial medical evidence.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8064852/

Record 798

pmcid: PMC8610925

pmid: 34877345

doi: 10.12998/wjcc.v9.i32.10018

title: Rare mutation in MKRN3 in two twin sisters with central precocious puberty: Two case reports.

authors

- Jiang LQ
- Zhou YQ
- Yuan K
- Zhu JF
- Fang YL
- Wang CL

journal: World journal of clinical cases

journalAbbr: World J Clin Cases

year: 2021

pubdate: 2021 Nov 16

volume: 9

issue: 32

pages: 10018-10023

abstract: BACKGROUND: Caused by premature activation of the hypothalamic-pituitary-gonadal axis, there is increasing incidence of central precocious puberty (CPP), especially in girls. Makorin ring finger protein 3 (MKRN3), a maternal imprinted gene with a highly conserved sequence, is the most common genetic etiology associated with CPP. Approximately 50 different mutations in MKRN3 have been found in CPP. CASE

SUMMARY: This case report involves identical twin sisters presenting with premature thelarche at the age of 6 years. The left hand bone age of both patients revealed advanced age (9 years). Pelvic B ultrasound indicated enlargement of the ovaries. Luteinizing hormone (LH) releasing hormone testing confirmed CPP. Whole-exome sequencing detected the c.841C>T mutation in MKRN3 , leading to a single base substitution, in the twins. This mutation was inherited from the father and paternal grandmother. After 3 mo of treatment with a gonadotropin-releasing hormone analog, levels of LH, follicle-stimulating hormone, and estradiol in the proband's sister returned to normal levels. CONCLUSION: Here, we report a rare mutation (c.841C>T) in MKRN3 in identical twin sisters with CPP.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8610925/

Record 799

pmcid: PMC8544644

pmid: 34722027

doi: 10.7759/cureus.18215

title: Rare Papillary Breast Carcinoma Incidentally Discovered After Trauma-Induced Hematoma.

authors

- Li B
- Nguyen J
- Williams CA
- Cardenas K
- Pidhorecky I

journal: Cureus

journalAbbr: Cureus

year: 2021

pubdate: 2021 Sep

volume: 13

issue: 9

pages: e18215

abstract: Papillary carcinoma of the breast is rare, comprising only 0.5% incidence of all breast cancers. Clinically the disease presents in postmenopausal women as a painless breast lump with possible bloody nipple discharge. Prognosis is favorable due to its slow growth. We present a 61-year-old woman incidentally diagnosed with papillary breast carcinoma after presenting with a trauma-induced hematoma of the right breast. The patient presented to our surgery oncology clinic for persistent right breast swelling secondary to a fall, despite initial incision and drainage (I&D) six weeks prior. She had no history of breast cancer. On presentation, her right breast was distended demonstrating an approximately 20cm ill-defined solid mass with skin changes consistent with a tense hematoma. CT scan demonstrated a large complex cystic and solid breast mass measuring 15.2cmx11.8cmx15.2cm with irregular peripheral solid hyperdense polypoid components. She then underwent a right breast incisional biopsy and hematoma evacuation. Frozen sections of the mass outer cavity wall and papillary projections were consistent with encapsulated papillary carcinoma (EPC). The patient was lost to follow-up and did not obtain definitive treatment. Breast cancer rarely presents as a breast hematoma. However, as in this case, if the hematoma fails to resolve, further investigation is warranted. The prognosis of EPC is excellent when identified and treated appropriately.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8544644/

Record 800

pmcid: PMC8033169

pmid: 33841329

doi: 10.3389/fendo.2021.625457

title: Real-World Effectiveness of Mineralocorticoid Receptor Antagonists in Primary Aldosteronism.

authors

- Tezuka Y
- Turcu AF

journal: Frontiers in endocrinology

journalAbbr: Front Endocrinol (Lausanne)

year: 2021

pubdate: 2021

volume: 12

issue:

pages: 625457

abstract: OBJECTIVE: To investigate how often target renin is pursued and achieved in patients with primary aldosteronism (PA) and other low renin hypertension (LRH) treated with mineralocorticoid receptor antagonists (MRAs), as reversal of renin suppression was shown to circumvent the enhanced cardiovascular and renal morbidity and mortality in these patients. PATIENTS AND METHODS: We conducted a retrospective cohort study of patients with PA and LRH treated with MRAs in an academic outpatient practice from January 1, 2000, through May 31, 2020. RESULTS: Of 30,777 patients with hypertension treated with MRAs, only 7.3% were evaluated for PA. 163 patients (123 with PA) had renin followed after MRA initiation. After a median follow-up of 124 [interquartile range, 65-335] days, 70 patients (43%) no longer had renin suppression at the last visit. The proportion of those who achieved target renin was higher in LRH than in PA (53% vs. 40%). Lower baseline serum potassium, lower MRA doses, and beta-blocker use were independently associated with lower odds of achieving target renin in PA, while male sex was associated with target renin in LRH. Overall, 50 patients (30.7%) had 55 adverse events, all from spironolactone, and 26 patients (52%) were switched to eplerenone or had a spironolactone dose reduction. CONCLUSION: Despite evidence that reversal of renin suppression confers cardio-renal protection in patients with PA and LRH, renin targets are followed in very few and are achieved in under half of such patients seen in an academic setting, with possibly even lower rates in community practices.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8033169/

Record 801

pmcid: PMC8716396

pmid: 34977018

doi: 10.3389/fcell.2021.767362

title: Recent Developments in Extracellular Matrix Remodeling for Fat Grafting.

authors

- Bi X
- Li Y
- Dong Z
- Zhao J
- Wu W
- Zou J
- Guo L
- Lu F
- Gao J

journal: Frontiers in cell and developmental biology

journalAbbr: Front Cell Dev Biol

year: 2021

pubdate: 2021

volume: 9

issue:

pages: 767362

abstract: Remodeling of the extracellular matrix (ECM), which provides structural and biochemical support for surrounding cells, is vital for adipose tissue regeneration after autologous fat grafting. Rapid and high-quality ECM remodeling can improve the retention rate after fat grafting by promoting neovascularization, regulating stem cells differentiation, and suppressing chronic inflammation. The degradation and deposition of ECM are regulated by various factors, including hypoxia, blood supply, inflammation, and stem cells. By contrast, ECM remodeling alters these regulatory factors, resulting in a dynamic relationship between them. Although researchers have attempted to identify the cellular sources of factors associated with tissue regeneration and regulation of the microenvironment, the factors and mechanisms that affect adipose tissue ECM remodeling remain incompletely understood. This review describes the process of adipose ECM remodeling after grafting and summarizes the factors that affect ECM reconstruction. Also, this review provides an overview of the clinical methods to avoid poor ECM remodeling. These findings may provide new ideas for improving the retention of adipose tissue after fat transplantation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8716396/

Record 802

pmcid: PMC8186996

pmid: 34121945

doi: 10.1055/s-0041-1727272

title: Revision Breast Augmentation.

authors

- Denney BD
- Cohn AB
- Bosworth JW
- Kumbla PA

journal: Seminars in plastic surgery

journalAbbr: Semin Plast Surg

year: 2021

pubdate: 2021 May

volume: 35

issue: 2

pages: 98-109

abstract: Breast augmentation is consistently one of the most commonly performed aesthetic operations every year. Unfortunately, revision rates following primary augmentation remain as high as 36%. There are several causes for revision breast augmentation; however, the most common and challenging of these include capsular contracture, implant malposition, and ptosis of the aging breast following augmentation. Successful management of these problems requires knowledge on how to best treat the implant and capsule with the corresponding soft tissue simultaneously. While surgical management is important, understanding the pathological causes of these entities during the primary operation can reduce the need for revision. This article

utilizes the most up-to-date literature to review the appropriate clinical evaluation and surgical management of these complex cases.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8186996/

Record 803

pmcid: PMC8120345

pmid: 34007451

doi: 10.3892/br.2021.1434

title: Role of Critical Care Units in the management of obstetric patients (Review).

authors

- Koukoubanis K
- Prodromidou A
- Stamatakis E
- Valsamidis D
- Thomakos N

journal: Biomedical reports

journalAbbr: Biomed Rep

year: 2021

pubdate: 2021 Jul

volume: 15

issue: 1

pages: 58

abstract: Pregnancy is considered a natural process for the majority of women. However, a limited proportion of pregnancies and deliveries can present with a broad variety of complications that may require admission to a Critical Care Unit (CCU). In the present review, the indications of admission of obstetrical and postpartum patients to CCUs were critically evaluated with a particular focus on the management of their complications. The management of critically ill obstetric patients remains challenging due to the physiological changes that occur during pregnancy, pregnancy-related diseases and the need to carefully consider the well-being of the fetus before any intervention can be recommended/performed. Indications for admission to CCUs include both obstetric and non-obstetric conditions that may require continuous monitoring and further interventions. Hypertensive disorders of pregnancy and mass hemorrhage are amongst the most common causes of admission to CCUs in pregnant and postpartum women. The establishment of a diagnostic and care algorithm based on the contribution of a multidisciplinary team is of critical importance to aid in the determination of which patients will require intensive care, and to assist in deciding what type of critical care each critically ill patients receives.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8120345/

Record 804

pmcid: PMC8792128

pmid: 35111440

doi: 10.7759/cureus.20752

title: Role of MRI in Breast Cancer Staging: A Case-Based Review.

authors

- Sharma S

- Vicenty-Latorre FG
- Elsherif S
- Sharma S

journal: Cureus

journalAbbr: Cureus

year: 2021

pubdate: 2021 Dec

volume: 13

issue: 12

pages: e20752

abstract: Breast cancer is the most commonly diagnosed cancer in women in the United States and is also the second leading cause of cancer deaths in women. The five-year survival rate and overall prognosis are largely dependent on the stage at diagnosis. Our article highlights the role of magnetic resonance imaging (MRI) in breast cancer staging and updates to the American Joint Committee on Cancer (AJCC) 8th edition guidelines regarding breast cancer staging that are relevant to radiologists. It provides a case-based approach to emphasize the key findings that the radiologist should report on a breast MRI to aid the clinicians in staging and management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8792128/

Record 805

pmcid: PMC8318999

pmid: 34336074

doi: 10.1016/j.radcr.2021.06.057

title: Role of ultrasound in diagnosis of neonatal breast enlargement: a newborn case report.

authors

- D'Auria D
- Ferrara D
- Aragione N
- De Chiara C
- Argenziano G
- Noschese I
- Noviello D
- Esposito F

journal: Radiology case reports

journalAbbr: Radiol Case Rep

year: 2021

pubdate: 2021 Sep

volume: 16

issue: 9

pages: 2692-2696

abstract: Neonatal breast enlargement is a hormone-related condition, mostly asymptomatic\physiological, with a well-recognizable sonographic appearance but limited data in the literature. It can be uni-or bilateral. Typically described in the first week of life, the transient lesion disappears spontaneously within 6 months. The main

differential diagnosis is neonatal mastitis, a breast tissue infection that requires a specific antibiotic therapy. Knowledge of clinical and imaging findings is crucial, and ultrasound represents a reliable tool which allows a quick identification and an excellent examination of neonatal breast abnormalities. We aim to highlight sonographic classic features of benign neonatal breast enlargement showing how ultrasound rules out real breast masses helping to differentiate between it and mastitis when clinical and laboratory data are inconclusive. We describe a 2-day-old male with bilateral breast swelling more pronounced on the right side.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8318999/

Record 806

pmcid: PMC8753789

pmid: 34057841

doi: 10.1177/03000605211020227

title: Self-consciousness and depression in precocious pubertal children.

authors

- Huang H
- Liu L
- Su S
- Xie D

journal: The Journal of international medical research

journalAbbr: J Int Med Res

year: 2021

pubdate: 2021 May

volume: 49

issue: 5

pages: 3000605211020227

abstract: OBJECTIVE: To explore self-consciousness and depression in children with precocious puberty (PP) and analyse its effect on children of both sexes. METHODS: Sixty children with PP and 60 non-PP children matched for sex and age participated in the study. Children were assessed using the Birleson Depression Self-Rating Scale for Children and the Piers-Harris Children's Self-Concept Scale. RESULTS: There were significant differences in physical appearance and attributes, anxiety, happiness and satisfaction between PP children and non-PP children. PP children had significantly higher depression than non-PP children. In the PP group, girls were significantly more prone to anxiety and unhappiness than boys. Conclusion: There were sex differences in the effect of PP on children's self-consciousness, and girls were more prone to anxiety and unhappiness. More attention should be paid to improving the physical and mental health of children with PP.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8753789/

Record 807

pmcid: PMC8220097

pmid: 34189231

doi: 10.1016/j.jpra.2021.04.011

title: Severe Gestational Gigantomastia: from Mastectomy to Staged Autologous Breast Reconstruction. A Case Report.

authors

- Hassayoune N
- Mhallem Gziri M

- Lentini A
- Chrelias T
- Hammer J
- Berlière M
- Lengelé B
- Coyette M

journal: JPRAS open

journalAbbr: JPRAS Open

year: 2021

pubdate: 2021 Sep

volume: 29

issue:

pages: 65-70

abstract: Gestational gigantomastia (GGM) is a rare condition characterized by a massive overgrowth of breast tissue during pregnancy. Surgical sanction may be required when conservative measures fail. In this study, we report the case of a 29-year-old woman who presented with an evolutive GGM responsible for physical and emotional distress, despite medical treatment. A multidisciplinary decision was made to induce delivery at 32 weeks. In the postdelivery period, the patient developed breast wounds, complicated with septic cardiomyopathy. An emergency bilateral mastectomy was then carried out, together with banking of both nipple-areola complexes. Thereafter, delayed bilateral 2-stage breast reconstruction was started at 12 months with subcutaneous tissue expanders, later on followed by implants removal and autologous reconstruction with bilateral deep inferior epigastric artery perforator flaps and bilateral nipple replantation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8220097/

Record 808

pmcid: PMC10694696

pmid: 38084233

doi: 10.5114/hpr.2021.102349

title: Short Sexual Well-Being Scale - a cross-sectional validation among transgender and cisgender people.

authors

- Gerymski R

journal: Health psychology report

journalAbbr: Health Psychol Rep

year: 2021

pubdate: 2021

volume: 9

issue: 3

pages: 276-287

abstract: BACKGROUND: Sexual well-being is often mistakenly operationalized simply as sexual satisfaction. The concept of sexual well-being has been often used as an umbrella term for positive aspects of sexuality, apart from its negative spheres. A new concept of sexual well-being was proposed in line with the World Health Organization (WHO) definition of sexual health. The definition tried to enrich the concept of sexual well-being, but it did not solve the problem of its operationalization. The aim of this study was to verify the psychometric properties of the new scale measuring the concept of sexual well-being. PARTICIPANTS AND PROCEDURE:

Two separate studies were conducted for the psychometric validation of the Short Sexual Well-Being Scale (SSWBS). This was because the first study on a group of transgender people was characterized by a smaller than recommended sample for confirmatory factor analysis. On this basis, a second study was conducted on a group of cisgender people. The study on a sample of transgender people (N = 111) verified the psychometric properties of the 4-point version of the scale. In the study on the cisgender sample (N = 211), it was decided to verify the psychometric properties of the 7-point version of the scale and verify whether a bigger response scale would have a positive effect on the reliability of the questionnaire. RESULTS: The confirmatory factor analysis, reliability analysis using alpha and total omega coefficients, and two measures of validity (content and criterion validity) were applied. The results indicated that the SSWBS had good psychometric properties. CONCLUSIONS: Overall, the presented scale shows good psychometric properties. Both versions of the scale can be used in the quantitative research, but it is recommended to use the 7-point version of the scale. The SSWBS is one of the first questionnaires constructed in accordance with the sexual well-being concept.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10694696/

Record 809

pmcid: PMC7985562

pmid: 33778175

doi: 10.1016/j.tipsro.2021.03.001

title: Side effects in breast implants related to radiotherapy in breast cancer reconstructive surgery.

authors

- de Faria Castro Fleury E
- Jasmin Huanca Bernal K
- Lucena Miranda Madeiro A
- Luis Cervera Ocana W
- Carlos Vendramini Fleury J
- Caobianco L

journal: Technical innovations & patient support in radiation oncology

journalAbbr: Tech Innov Patient Support Radiat Oncol

year: 2021

pubdate: 2021 Jun

volume: 18

issue:

pages: 8-11

abstract: Breast reconstructive surgery with silicone implants is routinely one of the techniques performed immediately after a mastectomy and before adjuvant radiotherapy. Implant shell degradation may result in gel bleeding that can trigger capsular disease. The silicone corpuscle in contact with the implant fibrous capsule can promote an inflammatory reaction, identified as silicone-induced granuloma which is, related to clinical complaints referred to as breast implant illness. This short communication aims to demonstrate and discuss the impact of radiotherapy's side effects on patients with post-mastectomy reconstructive breast surgery with silicone implants followed by adjuvant radiation therapy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7985562/

Record 810

pmcid: PMC8307199

pmid: 34300843

doi: 10.3390/ma14143917

title: Silicone Breast Implant Coated with Triamcinolone Inhibited Breast-Implant-Induced Fibrosis in a Porcine Model.

authors

- Nam SY
- Ji HB
- Shin BH
- Chien PN
- Donmez N
- Zhang XR
- Huh BK
- Kim MJ
- Choy YB
- Heo CY

journal: Materials (Basel, Switzerland)

journalAbbr: Materials (Basel)

year: 2021

pubdate: 2021 Jul 14

volume: 14

issue: 14

pages:

abstract: Cosmetic silicone implants for breast reconstruction often lead to medical complications, such as abnormally excessive fibrosis driven by foreign body granulomatous inflammation. The purpose of this study was to develop a silicone breast implant capable of local and controlled release of a glucocorticoid drug triamcinolone acetonide (TA) for the prevention of silicone-breast-implant-induced fibrosis in a Yorkshire pig model (in vivo). Implants were dip-coated in a TA solution to load 1.85 μg/cm 2 of TA in the implant shell, which could release the drug in a sustained manner for over 50 days. Immunohistochemical analysis for 12 weeks showed a decline in tumor necrosis factor-α expression, capsule thickness, and collagen density by 82.2%, 55.2%, and 32.3%, respectively. Furthermore, the counts of fibroblasts, macrophages, and myofibroblasts in the TA-coated implants were drastically reduced by 57.78%, 48.8%, and 64.02%, respectively. The TA-coated implants also lowered the expression of vimentin and α-smooth muscle actin proteins, the major profibrotic fibroblast and myofibroblast markers, respectively. Our findings suggest that TA-coated silicone breast implants can be a promising strategy for safely preventing fibrosis around the implants.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8307199/

Record 811

pmcid: PMC8395279

pmid: 34446504

doi: 10.1136/bmjopen-2021-053876

title: Spironolactone for adult female acne (SAFA): protocol for a double-blind, placebo-controlled, phase III randomised study of spironolactone as systemic therapy for acne in adult women.

authors

- Renz S
- Chinnery F
- Stuart B

- Day L
- Muller I
- Soulsby I
- Nuttall J
- Thomas K
- Thomas KS
- Sach T
- Stanton L
- Ridd MJ
- Francis N
- Little P
- Eminton Z
- Griffiths G
- Layton AM
- Santer M

journal: BMJ open

journalAbbr: BMJ Open

year: 2021

pubdate: 2021 Aug 26

volume: 11

issue: 8

pages: e053876

abstract: INTRODUCTION: Acne is one of the most common inflammatory skin diseases worldwide and can have significant psychosocial impact and cause permanent scarring. Spironolactone, a potassium-sparing diuretic, has antiandrogenic properties, potentially reducing sebum production and hyperkeratinisation in acne-prone follicles. Dermatologists have prescribed spironolactone for acne in women for over 30 years, but robust clinical study data are lacking. This study seeks to evaluate whether spironolactone is clinically effective and cost-effective in treating acne in women. METHODS AND ANALYSIS: Women (≥18 years) with persistent facial acne requiring systemic therapy are randomised to receive one tablet per day of 50 mg spironolactone or a matched placebo until week 6, increasing to up to two tablets per day (total of 100 mg spironolactone or matched placebo) until week 24, along with usual topical therapy if desired. Study treatment stops at week 24; participants are informed of their treatment allocation and enter an unblinded observational follow-up period for up to 6 months (up to week 52 after baseline). Primary outcome is the Acne-specific Quality of Life (Acne-QoL) symptom subscale score at week 12. Secondary outcomes include Acne-QoL total and subscales; participant acne self-assessment recorded on a 6-point Likert scale at 6, 12, 24 weeks and up to 52 weeks; Investigator's Global Assessment at weeks 6 and 12; cost and cost effectiveness are assessed over 24 weeks. Aiming to detect a group difference of 2 points on the Acne-QoL symptom subscale (SD 5.8, effect size 0.35), allowing for 20% loss to follow-up, gives a sample size of 398 participants. ETHICS AND DISSEMINATION: This protocol was approved by Wales Research Ethics Committee (18/WA/0420). Follow-up to be completed in early 2022. Findings will be disseminated to participants, peer-reviewed journals, networks and patient groups, on social media, on the study website and the Southampton Clinical Trials Unit website to maximise impact. TRIAL REGISTRATION NUMBER: ISRCTN12892056;Pre-results.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8395279/

Record 812

pmcid: PMC8185527

pmid: 33960324

doi: 10.1530/EDM-20-0156

title: Status epilepticus and diabetes ketoacidosis: uncommon clinical presentations of acromegaly.

authors

- Chamba NG
- Amour AA
- Sadiq AM
- Lyamuya TR
- Assey EV
- Sadiq AM
- Howlett WP

journal: Endocrinology, diabetes & metabolism case reports

journalAbbr: Endocrinol Diabetes Metab Case Rep

year: 2021

pubdate: 2021 Jan 1

volume: 2021

issue:

pages:

abstract: SUMMARY: Acromegaly is a rare disease caused by hypersecretion of the growth hormone (GH). Most cases are caused by either pituitary microadenoma or macroadenoma. The GH producing tumors present with clinical manifestations of acromegaly due to excessive GH secretion or symptoms resulting from mass effects of the enlarging tumor. The physical changes are usually slow and, therefore, recognition of the disease is delayed. These adenomas are never malignant but can have significant morbidity and mortality. A subgroup of patients with acromegaly present with severe hyperglycemia resulting in diabetic ketoacidosis (DKA) which requires insulin. Rarely are pituitary tumors responsible for generalized convulsions except when they are too large. We hereby present two cases, the first is that of a 26-year-old female who presented with new onset status epilepticus, DKA with a 1-year history of diabetes mellitus (DM). On examination, she had clinical features of acromegaly. The second case is that of a 34-year-old female who presented with new onset status epilepticus, hyperglycemia with a history of recently diagnosed DM, and features of gigantism. In both cases, their diagnosis was confirmed by elevated serum GH and later by elevated insulin-like growth factor type 1 levels, and CT of the head demonstrating large pituitary macroadenoma. The importance of clinical history and examination, as well as investigations is vital in the recognition of acromegaly. The prognosis of acromegalic patients depends on early clinical recognition and tumor size reduction by either medical or surgical therapy. LEARNING POINTS: Conditions such as status epilepticus and DKA may be clinical presentations in patients presenting with acromegaly. Seizures are rare in people with pituitary adenoma and typically occur when the tumor invades the suprasellar area due to mass effect on the brain. This article shows how best we were able to manage the acromegaly complications in a low resource setting. Hyperprolactinemia in acromegaly may be due to disruption of the normal dopaminergic inhibition of prolactin secretion due to mass effect of the macroadenoma, and around 25% of GH-secreting adenomas co-secrete prolactin.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8185527/

Record 813

pmcid: PMC8183742

pmid: 34087837

doi: 10.1097/MD.0000000000025962

title: Surgical management of complicated gynecomastia (associated with foreign body injection) with single-port 3-dimensional videoscope-assisted endoscopic subcutaneous mastectomy and concurrent liposuction: A case report.

authors

- Wen TC
- Lai HW
- Mok CW
- Chen ST
- Chen DR
- Kuo SJ

journal: Medicine

journalAbbr: Medicine (Baltimore)

year: 2021

pubdate: 2021 Jun 4

volume: 100

issue: 22

pages: e25962

abstract: RATIONALE: Gynecomastia is a common benign breast disorder in men. Surgical management of gynecomastia includes that of a subcutaneous mastectomy with or without concurrent liposuction. Herein, the authors presented a case of complicated gynecomastia (gynecomastia with concurrent foreign body injection) which was successfully managed with an innovative technique that offered acceptable operative time, minimal complications, good recovery and satisfactory aesthetic outcome. PATIENT CONCERNS: A 39-year-old Taiwanese man who developed gynecomastia along with self-injection of foreign body (salad oil) over the past 10 years for breast enlargement presented as symptomatic bilateral breast lumps. DIAGNOSIS: Bedside sonography revealed multiple large droplets of oil in the subcutaneous tissue bilaterally, resembling cystic lesions. INTERVENTION: Bilateral single-port 3-dimensional videoscope-assisted endoscopic subcutaneous mastectomy was performed after bilateral breast liposuction. Operative findings include bilateral gynecomastia and previous bilateral breast foreign body material. The total weight of lipoaspirate was 400 grams and 300 grams for right and left side respectively. Subcutaneous mastectomy specimen weight was 820 grams and 661 grams for right and left breast tissue. OUTCOMES: Operative duration was 315 minutes and intraoperative blood loss at 150 ml. Patient was discharged 2 days after the operation, and subsequent follow up ultrasound showed complete removal of foreign bodies and fibrotic breast tissue. Patient was satisfied with the post-operative aesthetic outcomes. LESSONS: Single-port 3-dimensional videoscope-assisted endoscopic subcutaneous mastectomy with concurrent liposuction is a promising and safe surgical option for patient with complicated gynecomastia and severe fibrosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8183742/

Record 814

pmcid: PMC8614637

pmid:

doi: 10.1002/14651858.CD014917

title: Systemic pharmacological treatments for acne: an overview of systematic reviews.

authors

- Costa CS
- Bagatin E
- Yang Z
- Pacheco RL
- Magin P
- Sá Urtiga Santos L

- Pereira T
- Riera R

journal: The Cochrane Database of Systematic Reviews

journalAbbr: Cochrane Database Syst Rev

year: 2021

pubdate: 2021

volume: 2021

issue: 11

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8614637/

Record 815

pmcid: PMC7982491

pmid: 33500232

doi: 10.1016/j.ijscr.2020.11.047

title: Temporary banking of the nipple-areola complex in breast reconstruction following mastectomy for gigantomastia.

authors

- Sowa Y
- Hori T
- Kodama T
- Numajiri T

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2021

pubdate: 2021 Mar

volume: 80

issue:

pages: 105297

abstract: INTRODUCTION: Gigantomastia is a breast disorder characterized by exaggerated rapid growth of the breasts, generally bilaterally. In some severe cases, mastectomy is required to ensure safe delivery or control disease progression or recurrence. Subsequently, most patients want to undergo breast reconstruction, including the nipple-areola complex (NAC). PRESENTATION OF CASE: Here, we report our experience with temporary banking of the NAC in a patient who underwent mastectomy for severe Gigantomastia. Each NAC was temporarily transplanted into the axilla as banking tissue for NAC reconstruction at a later date. Although the color of the NAC was slightly lighter after reconstruction, it mainly kept its original color and texture in addition to medical tattooing technique. At present, there has been no recurrence and the patient is fully satisfied with her appearance. DISCUSSION: In this case, mastectomy was recommended because of an unbearable breast size that disturb a safety delivery, as well as respiratory and cardiac complications and skin ulcer control. Because the disease is not pathologically malignant, temporary preservation of NAC allows it to be safely used again for later nipple reconstruction. CONCLUSION: Temporary banking of the nipple-areola complex in breast reconstruction following breast resection including NAC, would be one of good surgical options for benign breast tumors like gigantomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7982491/

Record 816

pmcid: PMC8090319

pmid:

doi: 10.1210/jendso/bvab048.325

title: The Development of Hypogonadotropic Hypogonadism After Starting Low Dose Mifepristone for Cushing's Syndrome.

authors

- Rowell JV

journal: Journal of the Endocrine Society

journalAbbr: J Endocr Soc

year: 2021

pubdate: 2021 May 3

volume: 5

issue: Suppl 1

pages: A160-1

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8090319/

Record 817

pmcid: PMC7932778

pmid: 33708258

doi: 10.1155/2021/8854182

title: The Effect of Jazar Supplement on Quality of Life and Sexual Function in Postmenopausal Women: A Double-Blind, Randomized, Placebo-Controlled Trial.

authors

- Hafizi S
- Abbassian A
- Tabarrai M

journal: Evidence-based complementary and alternative medicine : eCAM

journalAbbr: Evid Based Complement Alternat Med

year: 2021

pubdate: 2021

volume: 2021

issue:

pages: 8854182

abstract: BACKGROUND: Menopause is one of the most critical stages of a woman's life and is accompanied by symptoms including hot flashes, night sweats, mood swings, sexual dysfunction, mucus atrophy, and vaginal dryness. Women tend to use complementary and alternative medicine such as herbs rather than hormone replacement therapy to alleviate these symptoms. The purpose of this study is to examine the effects of Jazar supplement (herbal supplement comprising Vitex, fennel, and carrot seeds) on sexual function, quality of life,

and vaginal atrophy in postmenopausal women. METHODS: This study was a randomized clinical trial conducted on ninety postmenopausal women. Participants were randomized to intervention/control groups using block randomization. The intervention group received four Jazar capsules (500 mg each) daily, while the control group received a placebo for eight weeks. Data were gathered using a socio-demographic questionnaire, the Female Sexual Function Index (FSFI), and the Menopause-Specific Quality of Life (MENQOL) before the intervention and at weeks 4, 8, and 10. Vaginal pH and vaginal maturation index (VMI) were measured before and at the end of the study. RESULTS: Eighty-four women completed the trial, and six women withdrew. There were no remarkable differences between the two groups in terms of quality of life, sexual function, VMI, and vaginal pH at baseline. At the end of the study, participants in the intervention group had a significantly lower score in terms of quality of life ($P < 0.001$) and vaginal pH ($P =0.001$), and a higher FSFI ($P < 0.001$) and VMI ($P =0.030$) score compared to the control group. CONCLUSION: Based on the findings of the present study on menopausal women, the Jazar supplement significantly improved quality of life and sexual function and prevented or delayed vaginal atrophy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7932778/

Record 818

pmcid: PMC9580767

pmid: 36303983

doi: 10.3389/frph.2021.629673

title: The Impact and Management of Gynaecomastia in Klinefelter Syndrome.

authors

- Raheem AA
- Zaghloul AS
- Sadek AMG
- Rayes B
- Abdel-Raheem TM

journal: Frontiers in reproductive health

journalAbbr: Front Reprod Health

year: 2021

pubdate: 2021

volume: 3

issue:

pages: 629673

abstract: Gynecomastia is defined as a palpable enlargement of the male breast, secondary to an increase in the glandular and stromal breast tissue. Gynecomastia is encountered in up to 80% of Klinefelter syndrome cases. The pathophysiology involves testosterone/estrogen imbalance. This review article will further explore the pathophysiology of gynecomastia along with the different lines of management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9580767/

Record 819

pmcid: PMC8568796

pmid: 34744944

doi: 10.3389/fpsyg.2021.765315

title: The Psychosocial Consequences of Prostate Cancer Treatments on Body Image, Sexuality, and Relationships.

authors

- Mainwaring JM
- Walker LM
- Robinson JW
- Wassersug RJ
- Wibowo E

journal: Frontiers in psychology

journalAbbr: Front Psychol

year: 2021

pubdate: 2021

volume: 12

issue:

pages: 765315

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8568796/

Record 820

pmcid: PMC8477287

pmid: 34558448

doi: 10.4103/aam.aam_41_20

title: The relationship between body weight and dietary habits with respect to the timing of puberty among saudi children and adolescents.

authors

- Shahatah MA
- Jadkarim AM
- Banjar RZ
- Kabli YO
- Milyani AA
- Al-Agha AE

journal: Annals of African medicine

journalAbbr: Ann Afr Med

year: 2021

pubdate: 2021 Jul-Sep

volume: 20

issue: 3

pages: 193-197

abstract: OBJECTIVE: The objective is to investigate the timing of pubertal onset as determined by the development of secondary sexual characteristics in relation to body weight and dietary patterns among Saudi females. METHODS: Children/adolescents visiting general and endocrinology pediatric clinics in King AbdulAziz University Hospital were invited to participate in this study. Female subjects between the ages of 5 and 20 years were included in this study, and those with syndromic disease, chronic comorbidities, endocrinopathies, organic causes of precocious puberty, positive family history of early pubertal onset, and under chronic medication were

excluded from the study. Data were collected through clinical interviews with the consent of the legal guardians, and physical examinations were conducted. RESULTS: A. UNLABELLED: total of 164 females were investigated. The mean age of thelarche, adrenarche, and menarche was 10, 11.3, and 12.2 years, respectively. We found a significant correlation between higher weight standard deviation and an earlier age of both thelarche and adrenarche. In addition, daily consumption of fast foods was significantly associated with an earlier menarchal age. Consumption of nonorganic poultry was linked to early thelarche. CONCLUSION: Efforts should be directed to increase public and community awareness that fast food consumption, inorganic poultry, and higher body weight are important modifiable factors that lead to an earlier onset of female puberty across different parameters: breast development, adrenarche, and menstruation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8477287/

Record 821

pmcid: PMC8342181

pmid: 34367709

doi: 10.1155/2021/9305811

title: The Role of Core Biopsy versus Vacuum-Assisted Breast Biopsy In Primary Breast Angiosarcoma.

authors

- Abate A
- Querques G
- Giovanazzi R
- Di Bella C
- Besostri V
- Gisabella M
- Maino C
- Ippolito D
- Corso R

journal: Case reports in radiology

journalAbbr: Case Rep Radiol

year: 2021

pubdate: 2021

volume: 2021

issue:

pages: 9305811

abstract: We report the case of a 45-year-old woman with a slow-growing palpable nodule on the left breast, confirmed as a well-defined opacity on mammography, corresponding to a 5 cm hyperechoic lesion on ultrasound, and considered, on the basis of clinical examination and radiological findings, to be consistent with a lipoma. One year later, the patient represented with an enlarged left breast mass and underwent further imaging investigation with subsequent diagnosis of primary breast angiosarcoma obtained via a Vacuum-Assisted Breast Biopsy. The patient developed metastatic disease and succumbed to the disease one year after definitive diagnosis. Primary breast angiosarcoma is a rare malignant vascular neoplasia, characterized by aggressive patterns, poor prognosis, and absence of pathognomonic radiological features. Currently, there are no evidence-based guidelines regarding treatment, even though wide surgical resection followed by chemo- and radiotherapy appears to improve survival.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8342181/

Record 822

pmcid: PMC7909172

pmid: 33663116

doi: 10.1097/MD.0000000000024882

title: The safety and efficacy of low-dose mineralocorticoid receptor antagonists in dialysis patients: A meta-analysis.

authors

- Zhu Y
- Liu Y
- Cai R
- Zheng D
- Liang X
- Tao M
- Jin J
- Li Y
- He Q

journal: Medicine

journalAbbr: Medicine (Baltimore)

year: 2021

pubdate: 2021 Feb 26

volume: 100

issue: 8

pages: e24882

abstract: INTRODUCTION: Our aim was to evaluate the safety and efficacy of low-dose mineralocorticoid receptor antagonists (MRAs) in dialysis patients. METHODS: We systematically searched PubMed, EMBASE, and Cochrane libraries for clinical trials on the use of MRAs in dialysis patients. Review Manager 5.3 software was used to analyze relevant data and evaluate the quality of evidence. RESULTS: We identified nine randomized controlled trials including 1128 chronic dialysis patients. In terms of safety, when hyperkalemia was defined as serum potassium level ≥5.5 mmol/L, low-dose MRAs were significantly associated with hyperkalemia (relative risk [RR] 1.76, 95% confidence intervals [CI] 1.07-2.89, P = .02); however, when hyperkalemia was defined as serum potassium level ≥6.0 mmol/L or serum potassium level ≥6.5 mmol/L, no significant association was observed between low-dose MRAs and hyperkalemia (RR 1.40, 95% CI 0.83-2.37, P = .20; RR 1.98, 95% CI 0.91-4.30, P = .09, respectively). Use of low-dose MRAs can reduce cardiovascular mortality by 54% compared with the control group (0.46, 95% CI 0.28-0.76, P = .003). Similarly, the RR of all-cause mortality for the low-dose MRAs group was 0.48 (95% CI 0.33-0.72, P = .0003). CONCLUSION: Low-dose MRAs may benefit dialysis patients without significantly increasing moderate to severe hyperkalemia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7909172/

Record 823

pmcid: PMC8231352

pmid: 34165371

doi: 10.1080/13880209.2021.1936078

title: Traditional medicines and their common uses in central region of Syria: Hama and Homs - an ethnomedicinal survey.

authors

- Khatib C
- Nattouf A
- Hasan Agha MI

journal: Pharmaceutical biology

journalAbbr: Pharm Biol

year: 2021

pubdate: 2021 Dec

volume: 59

issue: 1

pages: 778-788

abstract: CONTEXT: Since ancient times, traditional Arabic medicine (TAM) has been used to treat various diseases in Syria. They are cost-effective with fewer side effects and are more suitable for long-term use compared with chemically synthesized medicines. In addition, the scientific importance is manifested, as this survey proceeds, for the purposes of verifying and documenting these traditional medicines and their common uses. OBJECTIVE: We conducted ethnobotanical and ethnomedicine research on plants traditionally used to treat various diseases in central region of Syria. METHODS: Information was collected from 2019 to 2021 from the cities of Homs and Hama and their villages, which are two governorates located in central Syria, after interviews with traditional practitioners called Attarin , and many other people. Plant specimens were collected according to different references concerning medicinal plants of Syria, to document the traditional uses of plants at least two of the traditional healers and three other people were asked. RESULTS: In this survey, we listed 76 medicinal plants belonging to 39 families in alphabetical order with the parts used and the method of preparation according to their therapeutic use, which are used to treat 106 ailments. CONCLUSIONS: Many of the uses of medicinal plants mentioned in this survey are still under study. There is no doubt that this study will provide new data that could contribute to further pharmacological discoveries by identifying the active ingredients and their mechanism of effect by doing additional pharmacological work to confirm the alleged biological activities of these plants.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8231352/

Record 824

pmcid: PMC8194428

pmid: 33942761

doi: 10.4103/lungindia.lungindia_336_20

title: Tuberculous cold abscess of the chest wall masquerading as unilateral apparent gynecomastia.

authors

- Divya R
- Rajesh V
- Augustine J
- Cleetus M

journal: Lung India : official organ of Indian Chest Society

journalAbbr: Lung India

year: 2021

pubdate: 2021 May-Jun

volume: 38

issue: 3

pages: 289-292

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8194428/

Record 825

pmcid: PMC8534015

pmid: 34680362

doi: 10.3390/cancers13205217

title: Uncommon Variants of Mature T-Cell Lymphomas (MTCLs): Imaging and Histopathologic and Clinical Features with Updates from the Fourth Edition of the World Health Organization (WHO) Classification of Lymphoid Neoplasms.

authors

- Salem AE
- Zaki YH
- El-Hussieny G
- ElNoueam KI
- Shaaban AM
- Koppula BR
- Yang M
- Salama M
- Elsayes KM
- Covington MF

journal: Cancers

journalAbbr: Cancers (Basel)

year: 2021

pubdate: 2021 Oct 18

volume: 13

issue: 20

pages:

abstract: Understanding the pathogenesis and molecular biology of malignant lymphomas is challenging, given the complex nature and incongruity of these disorders. The classification of lymphoma is continually evolving to account for advances in clinical, pathological, molecular biology and cytogenetic aspects, which impact our understanding of these disorders. The latest fourth edition of the WHO classification of lymphoid malignancies was released in 2016 to account for these changes. Additionally, unlike B-cell lymphomas (BCL), T-cell lymphomas (TCL) are uncommon, and may be sporadically experienced in clinical practice. These disorders are rare, thus early diagnosis is challenging for both physicians and radiologists, owing to the overlap in clinical and imaging features with other, more common disorders. We aim to discuss some rare variants of T-cell lymphomas, including clinicopathologic and imaging features, as well as to give a glimpse of the updates contained within the new 2016 WHO classification.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8534015/

Record 826

pmcid: PMC8575233

pmid: 34759477

doi: 10.4103/ijcm.IJCM_429_20

title: Understanding Social Media Usage and Engagement among Women to inform Breast Cancer Knowledge and Prevention Practices: Cross - sectional study in Delhi -National Capital Region of India.

authors

- Sinha N
- Sharma A

journal: Indian journal of community medicine : official publication of Indian Association of Preventive & Social Medicine

journalAbbr: Indian J Community Med

year: 2021

pubdate: 2021 Jul-Sep

volume: 46

issue: 3

pages: 411-415

abstract: BACKGROUND: Breast cancer is the major concern worldwide and in India too. Lack of awareness is one of the causes of increasing mortality rate in India. Social media is playing an important role in health communication including breast cancer information. In India, number of women are using social media. OBJECTIVE: To explore the impact of social media usage and engagement in enhancing knowledge and practices to prevent breast cancer among women of India. METHODOLOGY: A cross-sectional study was conducted in Delhi-National Capital Region of India with a sample of 649 women (response rate 83.51%). The questionnaire consisted of three sections. In first section, sociodemographic details (four items) were collected, second section contained five items on social media use and engagement and third part included items on knowledge about risk factors (seven items), symptoms (eight items), and screening (six items) of breast cancer and practice (seven items). Descriptive statistics, Chi-square, Cramer-V test, and structural equation modeling-Analysis of a moment structure were used to identify the relationship between social media engagement and knowledge and practices of women. RESULTS: Around 80% (431/542) of women have medium level of social media engagement and 20% are highly engaged. The slope coefficient of the relationship between social media engagement and knowledge is 0.805 and between knowledge and practice is 0.309, Chi-square value is 52.053 and 29.624, Cramer-V statistics is 0.310 and 0.165, respectively, which indicates significant relationship. CONCLUSION: The study result justified significant impact of social media engagement on knowledge and practices of women to prevent breast cancer.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8575233/

Record 827

pmcid: PMC7858550

pmid: 33552807

doi: 10.1097/GOX.0000000000003335

title: Unilateral Breast Inflation Caused by Breastfeeding after Polyacrylamide Hydrogel Injection.

authors

- Ebisudani S
- Inagawa K
- Suzuki Y
- Osugi I

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2021

pubdate: 2021 Jan

volume: 9

issue: 1

pages: e3335

abstract: Polyacrylamide hydrogel (PAAG) has been widely used in Russia and China as an injection material for cosmetic surgery. We report the case of a 36-year-old woman who noted breast enlargement subsequent to breastfeeding, after a PAAG injection. In 2016, a PAAG injection was administered to her under both mammary glands at a nearby cosmetic surgery clinic for breast augmentation. After she started breastfeeding following delivery in 2020, she was admitted to our hospital because of a rapid left breast enlargement and unbearable pain. Contrast-enhanced chest computed tomography revealed marked fluid retention in the left breast, and the mammary gland tissue had been pushed outward. A skin incision revealed a significant amount of yellowish-white odorless fluid accumulation. On surgery, a foreign body remained around the pectoralis major muscle, but complete removal was impossible. Currently, 3 months have passed since the operation; however, the patient has not experienced any further pain. There have been many reports on complications caused by PAAG injection. In our case, commencing breast feeding after receiving PAAG injections resulted in a rapid unilateral breast enlargement, and there is no similar report from Japan. In this case, suppressing lactation and rapid removal of the foreign body is the most important measure. It is difficult to completely remove PAAG once it has been injected. PAAG injections for breast augmentation should be avoided in all patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7858550/

Record 828

pmcid: PMC7806408

pmid: 32683812

doi: 10.3345/cep.2020.00521

title: Why should we be concerned about early menarche?

authors

- Lee HS

journal: Clinical and experimental pediatrics

journalAbbr: Clin Exp Pediatr

year: 2021

pubdate: 2021 Jan

volume: 64

issue: 1

pages: 26-27

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7806408/

Record 829

pmcid: PMC7825134

pmid: 33418872

doi: 10.3390/toxins13010035

title: Zearalenone and Its Metabolites-General Overview, Occurrence, and Toxicity.

authors

- Ropejko K

- Twarużek M

journal: Toxins

journalAbbr: Toxins (Basel)

year: 2021

pubdate: 2021 Jan 6

volume: 13

issue: 1

pages:

abstract: Mycotoxins are secondary metabolites of filamentous fungi and represent one of the most common groups of food contaminants with low molecular weight. These toxins are considered common and can affect the food chain at various stages of production, harvesting, storage and processing. Zearalenone is one of over 400 detected mycotoxins and produced by fungi of the genus Fusarium ; it mainly has estrogenic effects on various organisms. Contaminated products can lead to huge economic losses and pose risks to animals and humans. In this review, we systemize information on zearalenone and its major metabolites.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7825134/

Record 830

pmcid: PMC8800754

pmid: 32762164

doi: 10.3785/j.issn.1008-9292.2020.04.03

title: [Efficacy and safety of letrozole in treatment of McCune-Albright syndrome girls with peripheral precocious puberty].

authors

- Xu D
- Lu W
- Wang X
- Wang J
- Xie Y
- Dong Z
- Wang W

journal: Zhejiang da xue xue bao. Yi xue ban = Journal of Zhejiang University. Medical sciences

journalAbbr: Zhejiang Da Xue Xue Bao Yi Xue Ban

year: 2020

pubdate: 2020 May 25

volume: 49

issue: 3

pages: 291-296

abstract: OBJECTIVE: To evaluate the efficacy and safety of the third-generation aromatase inhibitor letrozole in the treatment of McCune-Albright syndrome (MAS) girls with peripheral precocious puberty. METHODS: Twenty-one MAS girls with peripheral precocious puberty treated in Pediatrics Department of Ruijin Hospital, Shanghai Jiao Tong University School of Medicine from March 2012 to June 2017 were enrolled in the study. Patients presented with repeated vaginal bleeding, premature breast enlargement, café-au-lait spots or dysplasia of bone fibers, and low levels of luteinizing hormone (LH) and follicle-stimulating hormone (FSH); and the congenital adrenal hyperplasia, estrogen-producing tumors, and exogenous estrogen intake were excluded.

Letrozole were administrated at a dose of 0.5-2 mg·m -2 ·d -1 for 6 to 12 months. The patients were observed for changes in breast staging, vaginal bleeding, sex hormone levels, liver function and bone age changes, and changes in uterine and ovarian volume. RESULTS: After treatment, bone age/chronological age (BA/CA)was decreased from 1.23±0.30 to 1.11±0.18 (P < 0.01); the predicted adult height (PAH) increased from (156.2±5.9)cm to (158.4±2.1)cm after treatment (P < 0.05); the vaginal bleeding was reduced and the estradiol level decreased, while the teststosterone level and the uterus showed no significant increase, and no adverse reactions such as ovarian torsion and abnormal liver function were observed. CONCLUSIONS: Precocious puberty is one of the most common endocrine manifestations in MAS. Our findings suggest that letrozole may be an effective and safe therapy to precocious puberty in girls with McCune-Albright Syndrome.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8800754/

Record 831

pmcid: PMC7398738

pmid: 32766014

doi: 10.7759/cureus.8972

title: A Case of Male Breast Cancer Patient with CHEK2*1100delC Mutation.

authors

- Nguyen QD
- Tavana A
- Saenz Rios F
- Posleman Monetto FE
- Robinson AS

journal: Cureus

journalAbbr: Cureus

year: 2020

pubdate: 2020 Jul 2

volume: 12

issue: 7

pages: e8972

abstract: Male breast cancer (MBC) is a rare disease that accounts for less than one percent of all breast cancers. The association between BRCA1 and BRCA2 mutations and MBC has been well-established; recent data suggest that CHEK2 1100delC heterozygosity is also associated with an increased risk of MBC. Herein, we present the case of a 47-year-old male who was initially diagnosed with bilateral symmetric gynecomastia on a diagnostic mammogram performed for right breast palpable lump. Sixteen months after his diagnosis of gynecomastia, he presented with enlarging right breast palpable lumps and underwent a diagnostic mammogram and breast ultrasound. Ultrasound-guided biopsies were performed on the right breast mass and axillary lymphadenopathy. Pathology revealed right breast invasive ductal carcinoma (IDC) and right axillary metastatic lymphadenopathy. Subsequent genetic testing found CHEK2*1100delC mutation. This case report focuses on the presentation, diagnosis, and management of breast cancer, as well as long-term cancer screening in the setting of CHEK2 mutation in a relatively young male patient.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7398738/

Record 832

pmcid: PMC7544397

pmid: 33133945

doi: 10.1097/GOX.0000000000003092

title: A Comparison of 28 Published Augmentation/Mastopexy Techniques Using Photographic Measurements.

authors

- Swanson E

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2020

pubdate: 2020 Sep

volume: 8

issue: 9

pages: e3092

abstract: BACKGROUND: Numerous augmentation/mastopexy methods have been described in the literature, including those reported in 16 publications in 2019. However, objective measurements of breast dimensions are lacking, leaving little information on which to base treatment selection. The goal is to increase upper pole projection using an implant and correct ptosis by elevating the lower pole with the mastopexy. METHODS: A PubMed search was conducted to identify published augmentation/mastopexy methods. Lateral photographs were matched for size and orientation and then compared using a 2-dimensional measurement system. Measurements were compared for 5 common approaches-vertical; periareolar; inverted-T, central mound; inverted-T, superior pedicle; and inverted-T, inferior pedicle. Four publications not fitting these 5 groups were also evaluated. Measurement parameters included breast projection, upper pole projection, lower pole level, breast mound elevation, nipple level, area, and breast parenchymal ratio. RESULTS: A total of 106 publications were identified; 32 publications included lateral photographs suitable for comparison. Twenty-eight publications fitting 1 of the 5 groups were compared. All published augmentation/mastopexy methods increased breast projection and upper pole projection, although not significantly for inverted-T methods. Vertical augmentation/mastopexy was the only method that significantly raised the lower pole level ($P < 0.05$). The vertical technique also significantly ($P < 0.01$) increased the breast parenchymal ratio. Periareolar; inverted-T, central mound; and inverted-T, inferior pedicle methods produced nonsignificant increments in the breast parenchymal ratio. CONCLUSIONS: Breast implants increase breast projection and upper pole projection. Only vertical augmentation/mastopexy significantly elevates the lower pole. This method also significantly increases the breast parenchymal ratio, achieving the surgical objectives.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7544397/

Record 833

pmcid: PMC7136759

pmid: 32274220

doi: 10.1155/2020/3974829

title: A Rare Case of Nonsyndromic Generalized Radiculomegaly with a Literature Review.

authors

- Al-Obaida M

journal: Case reports in dentistry

journalAbbr: Case Rep Dent

year: 2020

pubdate: 2020

volume: 2020

issue:

pages: 3974829

abstract: Radiculomegaly is a rare condition involving elongated tooth roots. This condition has significant clinical implications and has been associated with syndromes such as oculofaciocardiodental syndrome. However, only a few nonsyndromic cases of radiculomegaly have been reported. Here, we report a case of nonsyndromic radiculomegaly in a Saudi individual. A subsequent review of the literature suggests endodontic management modalities for individuals with the dental findings demonstrated in the present case.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7136759/

Record 834

pmcid: PMC7549847

pmid: 33062475

doi: 10.7759/cureus.10352

title: A Rare Case of Pseudocyesis in a Patient With Bipolar Disorder.

authors

- Espiridion ED
- Fleckenstein C
- Boyle P
- Oladunjoye AO

journal: Cureus

journalAbbr: Cureus

year: 2020

pubdate: 2020 Sep 10

volume: 12

issue: 9

pages: e10352

abstract: Pseudocyesis is a rare condition in which a person has a false belief of being pregnant, accompanied by objective signs and symptoms of pregnancy, despite not being pregnant. Confirmation of pseudocyesis is achieved with a negative result of beta-human chorionic gonadotropin in the blood and/or urine and negative ultrasound finding. Most cases of pseudocyesis occur in the setting of major depressive disorder or psychotic disorder, with very few occurring during a manic episode of bipolar disorder. Hence, we present a 30-year-old woman with pseudocyesis in the setting of bipolar disorder, specifically within a current manic episode with features of psychosis. The patient was found in the woods naked, with signs of psychosis. She described symptoms of increasing abdominal size, whitish discharge from her nipple, and feeling of fetal movement. The patient continued to believe she was pregnant due to her symptoms, despite negative pregnancy tests on multiple occasions. She has a history of bipolar disorder and post-traumatic stress disorder. Her examination showed an obese woman, with a non-distended abdomen and non-palpable uterus with no breast tenderness or enlargement. The patient was given olanzapine for her agitation and was subsequently stabilized with haloperidol and lorazepam. She was restarted on her home medications, including risperidone, oxcarbazepine, and topiramate. She was later committed involuntarily and transferred to a long-term psychiatry facility. Pseudocyesis is a rare condition often associated with other psychiatric comorbidities. Our patient's presentation highlights one of the few cases ever formally documented in a developed country, as most of the cases reported are found in developing countries. More studies, including case series and systematic reviews, need to be done to better understand this rare condition and its other variants.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7549847/

Record 835

pmcid: PMC7029290

pmid: 32089683

doi: 10.1155/2020/5386193

title: A Review on Sex Steroid Hormone Estrogen Receptors in Mammals and Fish.

authors

- Amenyogbe E
- Chen G
- Wang Z
- Lu X
- Lin M
- Lin AY

journal: International journal of endocrinology

journalAbbr: Int J Endocrinol

year: 2020

pubdate: 2020

volume: 2020

issue:

pages: 5386193

abstract: Steroid hormones play essential roles in the reproductive biology of vertebrates. Estrogen exercises its effect through estrogen receptors and is not only a female reproductive hormone but acts virtually in all vertebrates, including fish, and is involved in the physiological and pathological states in all males and females. Estrogen has been implicated in mandible conservation and circulatory and central nervous systems as well as the reproductive system. This review intended to understand the structure, function, binding affinities, and activations of estrogens and estrogen receptors and to discuss the understanding of the role of sex steroid hormone estrogen receptors in mammals and fish.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7029290/

Record 836

pmcid: PMC7708693

pmid: 33304571

doi: 10.1016/j.amsu.2020.11.049

title: A successful breastfeeding after vertical scar reduction mammaplasty with superior pedicle: A case report.

authors

- Seswandhana R
- Anzhari S
- Dachlan I

journal: Annals of medicine and surgery (2012)

journalAbbr: Ann Med Surg (Lond)

year: 2020

pubdate: 2020 Dec

volume: 60

issue:

pages: 600-603

abstract: INTRODUCTION: Most of patients undergo reduction mammaplasty for aesthetic or therapeutic

reasons without consider the effect on breastfeeding function. Vertical scar mammaplasty with superior pedicle is expected to be a breast reduction procedure that can keep maintain the function of breastfeeding. This is the first recorded report of breastfeeding after vertical scar reduction mammaplasty with superior pedicle in Indonesia. PRESENTATION OF CASE: A 23 years old woman presented to the outpatient clinic with enlargement of both breast for 3 years. Physical examination showed bilateral breast enlargement. No tenderness, nodules, nor axillary lymph node enlargement were found. The patient was managed with vertical scar mammaplasty with superior pedicle. The patient was followed up with complication of skin excess and scarring on the bilateral submammary folds. We performed excision and resection procedures to eliminate the skin excess and scars without further complications. The patient was married and gave birth to her first and second child after two and five years following mammaplasty. The patient was able to provide exclusive breastfeeding for both of her children. DISCUSSION: Vertical scar mammaplasty with superior pedicle surgery is a surgical technique that combines a superior pedicle for the areola and performs a central-inferior quadrant resection for breast reduction. It only takes the tissue and glands that are located in the lower quadrant and still maintains the surrounding tissue and glands. This technique also maintains the integrity of nipple-areola complex (NAC) which also important in the lactation process. CONCLUSION: Vertical scar mammaplasty with superior pedicle can be one of the superior techniques in breast reduction which can maintain the breastfeeding function thereby increasing patient satisfaction.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7708693/

Record 837

pmcid: PMC7171614

pmid: 32351604

doi: 10.1155/2020/8091846

title: Acupuncture Improving Early Sexual Development of Girls with Peripheral Precocious Puberty: A Prospective Cohort Study.

authors

- Liu L
- Wan N
- Sun H

journal: Evidence-based complementary and alternative medicine : eCAM

journalAbbr: Evid Based Complement Alternat Med

year: 2020

pubdate: 2020

volume: 2020

issue:

pages: 8091846

abstract: OBJECTIVE: To study primarily on the effect of acupuncture on breast Tanner stage, serum sex hormone level, and TCM symptom scores in girls with peripheral precocious puberty (PPP). METHODS: 19 eligible patients diagnosed with PPP received acupuncture intervention for 12 weeks, twice a week for 12 weeks, 24 sessions of acupuncture treatment in all, and then follow-up for 12 weeks. The primary outcome was the change in serum luteinizing hormone (LH) level, follicle-stimulating hormone (FSH) level, and breast Tanner stage at 12-week treatment and 12-week follow-up. Serum estradiol (E2) level and TCM symptom scores were also assessed. RESULTS: Nineteen patients with peripheral precocious puberty were treated with acupuncture. After the 12-week acupuncture intervention, the serum LH level, breast Tanner stage, and TCM symptom scores decreased significantly compared with baseline ($P < 0.05$); the serum FSH and E2 level did not change significantly after the 12-week treatment ($P > 0.05$). After the 12-week follow-up, breast Tanner stage and TCM symptom scores decreased significantly compared with baseline ($P < 0.05$), and there was no statistical difference between serum sex hormone (LH, FSH, and E2) level and baseline level ($P > 0.05$). During the period of acupuncture treatment, no side effects or serious adverse events occurred. CONCLUSIONS:

Acupuncture is effective in regulating the hormone level and controlling early development process. It may be a viable alternative to the treatment of peripheral precocious puberty in girls. However, further randomized controlled trials are needed.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7171614/

Record 838

pmcid: PMC7013159

pmid: 32072071

doi: 10.1016/j.conctc.2020.100537

title: Add-on spironolactone as antagonist of the NRG1-ERBB4 signaling pathway for the treatment of schizophrenia: Study design and methodology of a multicenter randomized, placebo-controlled trial.

authors

- Hasan A
- Roeh A
- Leucht S
- Langguth B
- Hansbauer M
- Oviedo-Salcedo T
- Kirchner SK
- Papazova I
- Löhrs L
- Wagner E
- Maurus I
- Strube W
- Rossner MJ
- Wehr MC
- Bauer I
- Heres S
- Leucht C
- Kreuzer PM
- Zimmermann S
- Schneider-Axmann T
- Görlitz T
- Karch S
- Egert-Schwender S
- Schossow B
- Rothe P
- Falkai P

journal: Contemporary clinical trials communications

journalAbbr: Contemp Clin Trials Commun

year: 2020

pubdate: 2020 Mar

volume: 17

issue:

pages: 100537

abstract: BACKGROUND: Preclinical studies recently showed that the mineralocorticoid antagonist spironolactone acts also as an antagonist of the NRG1-ERBB4 signaling pathway and improves schizophrenia-like behaviour in Nrg1 transgenic mouse model. As this signaling pathway is critically linked to the pathophysiology of schizophrenia, especially in the context of working-memory dysfunction, spironolactone may be a novel treatment option for patients with schizophrenia targeting cognitive impairments. AIMS: To evaluate whether spironolactone added to an ongoing antipsychotic treatment improves cognitive functioning in schizophrenia. METHODS: The add-on spironolactone for the treatment of schizophrenia trial (SPIRO-TREAT) is a multicenter randomized, placebo-controlled trial with three arms (spironolactone 100 mg, spironolactone 200 mg and placebo). Schizophrenia patients are treated for three weeks and then followed-up for additional nine weeks. As primary outcome, we investigate changes in working memory before and at the end of the intervention phase. We will randomize 90 patients. Eighty-one patients are intended to reach the primary endpoint measure at the end of the three-week intervention period. Secondary endpoints include other measures of cognition, psychopathology, safety measures and biological measures. CONCLUSIONS: SPIRO-TREAT is the first study evaluating the efficacy of the mineralocorticoid receptor antagonist spironolactone to improve cognitive impairments in schizophrenia patients targeting the NRG1-ERBB4 signaling pathway. With SPIRO-TREAT, we intend to investigate a novel treatment option for cognitive impairments in schizophrenia that goes beyond the established concepts of interfering with dopaminergic neurotransmission as key pathway in schizophrenia treatment. CLINICAL TRIAL REGISTRATION: International Clinical Trials Registry Platform: http://apps.who.int/trialsearch/Trial2.aspx?TrialID=EUCTR2014-001968-35-DE.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7013159/

Record 839

pmcid: PMC7079360

pmid: 32183872

doi: 10.1186/s13643-020-01296-8

title: Adverse drug reactions associated with amitriptyline - protocol for a systematic multiple-indication review and meta-analysis.

authors

- Brueckle MS
- Thomas ET
- Seide SE
- Pilz M
- Gonzalez-Gonzalez AI
- Nguyen TS
- Harder S
- Glasziou PP
- Gerlach FM
- Muth C

journal: Systematic reviews

journalAbbr: Syst Rev

year: 2020

pubdate: 2020 Mar 17

volume: 9

issue: 1

pages: 59

abstract: BACKGROUND: Unwanted anticholinergic effects are both underestimated and frequently overlooked. Failure to identify adverse drug reactions (ADRs) can lead to prescribing cascades and the unnecessary use of over-the-counter products. The objective of this systematic review and meta-analysis is to explore and quantify the frequency and severity of ADRs associated with amitriptyline vs. placebo in randomized controlled trials (RCTs) involving adults with any indication, as well as healthy individuals. METHODS: A systematic search in six electronic databases, forward/backward searches, manual searches, and searches for Food and Drug Administration (FDA) and European Medicines Agency (EMA) approval studies, will be performed. Placebo-controlled RCTs evaluating amitriptyline in any dosage, regardless of indication and without restrictions on the time and language of publication, will be included, as will healthy individuals. Studies of topical amitriptyline, combination therapies, or including < 100 participants, will be excluded. Two investigators will screen the studies independently, assess methodological quality, and extract data on design, population, intervention, and outcomes ((non-)anticholinergic ADRs, e.g., symptoms, test results, and adverse drug events (ADEs) such as falls). The primary outcome will be the frequency of anticholinergic ADRs as a binary outcome (absolute number of patients with/without anticholinergic ADRs) in amitriptyline vs. placebo groups. Anticholinergic ADRs will be defined by an experienced clinical pharmacologist, based on literature and data from Martindale: The Complete Drug Reference. Secondary outcomes will be frequency and severity of (non-)anticholinergic ADRs and ADEs. The information will be synthesized in meta-analyses and narratives. We intend to assess heterogeneity using meta-regression (for indication, outcome, and time points) and I 2 statistics. Binary outcomes will be expressed as odds ratios, and continuous outcomes as standardized mean differences. Effect measures will be provided using 95% confidence intervals. We plan sensitivity analyses to assess methodological quality, outcome reporting etc., and subgroup analyses on age, dosage, and duration of treatment. DISCUSSION: We will quantify the frequency of anticholinergic and other ADRs/ADEs in adults taking amitriptyline for any indication by comparing rates for amitriptyline vs. placebo, hence, preventing bias from disease symptoms and nocebo effects. As no standardized instrument exists to measure it, our overall estimate of anticholinergic ADRs may have limitations. SYSTEMATIC REVIEW REGISTRATION: Submitted to PROSPERO; assignment is in progress.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7079360/

Record 840

pmcid: PMC8094274

pmid: 33107592

doi: 10.1002/14651858.CD007004.pub4

title: Aldosterone antagonists in addition to renin angiotensin system antagonists for preventing the progression of chronic kidney disease.

authors

- Chung EY
- Ruospo M
- Natale P
- Bolignano D
- Navaneethan SD
- Palmer SC
- Strippoli GF

journal: The Cochrane database of systematic reviews

journalAbbr: Cochrane Database Syst Rev

year: 2020

pubdate: 2020 Oct 27

volume: 10

issue: 10

pages: CD007004

abstract: BACKGROUND: Treatment with angiotensin-converting enzyme inhibitors (ACEi) and angiotensin receptor blockers (ARB) is used to reduce proteinuria and retard the progression of chronic kidney disease (CKD). However, resolution of proteinuria may be incomplete with these therapies and the addition of an aldosterone antagonist may be added to further prevent progression of CKD. This is an update of a Cochrane review first published in 2009 and updated in 2014. OBJECTIVES: To evaluate the effects of aldosterone antagonists (selective (eplerenone), non-selective (spironolactone or canrenone), or non-steroidal mineralocorticoid antagonists (finerenone)) in adults who have CKD with proteinuria (nephrotic and non-nephrotic range) on: patient-centred endpoints including kidney failure (previously know as end-stage kidney disease (ESKD)), major cardiovascular events, and death (any cause); kidney function (proteinuria, estimated glomerular filtration rate (eGFR), and doubling of serum creatinine); blood pressure; and adverse events (including hyperkalaemia, acute kidney injury, and gynaecomastia). SEARCH METHODS: We searched the Cochrane Kidney and Transplant Register of Studies up to 13 January 2020 through contact with the Information Specialist using search terms relevant to this review. Studies in the Register are identified through searches of CENTRAL, MEDLINE, and EMBASE, conference proceedings, the International Clinical Trials Register (ICTRP) Search Portal, and ClinicalTrials.gov. SELECTION CRITERIA: We included randomised controlled trials (RCTs) and quasi-RCTs that compared aldosterone antagonists in combination with ACEi or ARB (or both) to other anti-hypertensive strategies or placebo in participants with proteinuric CKD. DATA COLLECTION AND ANALYSIS: Two authors independently assessed study quality and extracted data. Data were summarised using random effects meta-analysis. We expressed summary treatment estimates as a risk ratio (RR) for dichotomous outcomes and mean difference (MD) for continuous outcomes, or standardised mean difference (SMD) when different scales were used together with their 95% confidence interval (CI). Risk of bias were assessed using the Cochrane tool. Evidence certainty was evaluated using GRADE. MAIN RESULTS: Forty-four studies (5745 participants) were included. Risk of bias in the evaluated methodological domains were unclear or high risk in most studies. Adequate random sequence generation was present in 12 studies, allocation concealment in five studies, blinding of participant and investigators in 18 studies, blinding of outcome assessment in 15 studies, and complete outcome reporting in 24 studies. All studies comparing aldosterone antagonists to placebo or standard care were used in addition to an ACEi or ARB (or both). None of the studies were powered to detect differences in patient-level outcomes including kidney failure, major cardiovascular events or death. Aldosterone antagonists had uncertain effects on kidney failure (2 studies, 84 participants: RR 3.00, 95% CI 0.33 to 27.65, I^2 = 0%; very low certainty evidence), death (3 studies, 421 participants: RR 0.58, 95% CI 0.10 to 3.50, I^2 = 0%; low certainty evidence), and cardiovascular events (3 studies, 1067 participants: RR 0.95, 95% CI 0.26 to 3.56; I^2 = 42%; low certainty evidence) compared to placebo or standard care. Aldosterone antagonists may reduce protein excretion (14 studies, 1193 participants: SMD -0.51, 95% CI -0.82 to -0.20, I^2 = 82%; very low certainty evidence), eGFR (13 studies, 1165 participants, MD -3.00 mL/min/1.73 m^2, 95% CI -5.51 to -0.49, I^2 = 0%, low certainty evidence) and systolic blood pressure (14 studies, 911 participants: MD -4.98 mmHg, 95% CI -8.22 to -1.75, I^2 = 87%; very low certainty evidence) compared to placebo or standard care. Aldosterone antagonists probably increase the risk of hyperkalaemia (17 studies, 3001 participants: RR 2.17, 95% CI 1.47 to 3.22, I^2 = 0%; moderate certainty evidence), acute kidney injury (5 studies, 1446 participants: RR 2.04, 95% CI 1.05 to 3.97, I^2 = 0%; moderate certainty evidence), and gynaecomastia (4 studies, 281 participants: RR 5.14, 95% CI 1.14 to 23.23, I^2 = 0%; moderate certainty evidence) compared to placebo or standard care. Non-selective aldosterone antagonists plus ACEi or ARB had uncertain effects on protein excretion (2 studies, 139 participants: SMD -1.59, 95% CI -3.80 to 0.62, I^2 = 93%; very low certainty evidence) but may increase serum potassium (2 studies, 121 participants: MD 0.31 mEq/L, 95% CI 0.17 to 0.45, I^2 = 0%; low certainty evidence) compared to diuretics plus ACEi or ARB. Selective aldosterone antagonists may increase the risk of hyperkalaemia (2 studies, 500 participants: RR 1.62, 95% CI 0.66 to 3.95, I^2 = 0%; low certainty evidence) compared ACEi or ARB (or both). There were insufficient studies to perform meta-analyses for the comparison between non-selective aldosterone antagonists and calcium channel blockers, selective aldosterone antagonists plus ACEi or ARB (or both) and nitrate plus ACEi or ARB (or both), and non-steroidal mineralocorticoid antagonists and selective aldosterone antagonists. AUTHORS' CONCLUSIONS: The effects of aldosterone antagonists when added to ACEi or ARB (or both) on the risks of death, major cardiovascular events, and kidney failure in people with proteinuric CKD are uncertain. Aldosterone antagonists may reduce proteinuria, eGFR, and systolic blood pressure in adults who have mild to moderate CKD but may increase the risk of hyperkalaemia, acute kidney injury and gynaecomastia when added to ACEi and/or ARB.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8094274/

Record 841

pmcid: PMC7005977

pmid: 32024430

doi: 10.1177/1557988319898991

title: An Educational Program to Help Patients Manage Androgen Deprivation Therapy Side Effects: Feasibility, Acceptability, and Preliminary Outcomes.

authors

- Wibowo E
- Wassersug RJ
- Robinson JW
- Santos-Iglesias P
- Matthew A
- McLeod DL
- Walker LM

journal: American journal of men's health

journalAbbr: Am J Mens Health

year: 2020

pubdate: 2020 Jan-Feb

volume: 14

issue: 1

pages: 1557988319898991

abstract: Androgen deprivation therapy (ADT), a common treatment for prostate cancer, is associated with physical, psychological, and sexual side effects that reduce patients' quality of life. The authors designed an educational program to prepare patients for managing these side effects. This paper describes an implementation model for national dissemination of the program, testing its feasibility and acceptability at the institutional and patient level. Postprogram changes in patients' self-efficacy to manage side effects and side effect bother are also explored. Patients on or anticipating ADT enrolled in the educational program. Pre and post intervention questionnaires measured patient satisfaction with the program, side effect bother, and self-efficacy to manage ADT side effects. The ADT Educational Program was deemed feasible and acceptable. Five of six targeted sites successfully launched the program with sufficient patient enrolment. Patient attendees were highly satisfied. Self-efficacy, bother, and use of management strategies were interrelated. Lower bother was associated with increased self-efficacy and more use of management strategies, and increased bother was associated with lower self-efficacy and less use of management strategies. Based on pre-post scores, improvements in patients' self-efficacy to manage ADT side effects were also observed. Results demonstrate that this brief educational program is feasible and acceptable to patients and cancer care institutions. The program appears to promote self-efficacy and the uptake of ADT management strategies for ADT side effects. The results of this study support the program implementation and suggest that improvements in self-efficacy after program participation may help patients adapt to ADT side effects.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7005977/

Record 842

pmcid: PMC7023973

pmid: 32071576

doi: 10.1055/s-0039-1700958

title: Application of Fat Grafting in Cosmetic Breast Surgery.

authors

- Davis MJ
- Perdanasari AT
- Abu-Ghname A
- Gonzalez SR
- Chamata E
- Rammos CK
- Winocour SJ

journal: Seminars in plastic surgery

journalAbbr: Semin Plast Surg

year: 2020

pubdate: 2020 Feb

volume: 34

issue: 1

pages: 24-29

abstract: The role of fat grafting to the breasts has evolved in the recent past, gaining several new applications within both reconstructive and aesthetic surgery. Initially used for reconstructive purposes to fill lumpectomy defects or to correct residual contour deformities after breast reconstruction, it has since made its way into cosmetic breast surgery and has grown to encompass a wide variety of new indications. Fat grafting in aesthetic breast surgery may be performed as a form of primary autologous breast augmentation or as an adjunct to implant-based breast augmentation to disguise implant edges. It may also be used to provide added volume after explant surgery or to provide improvements in breast contour alongside mastopexy techniques. In this article, we will review the current applications of fat grafting in aesthetic breast surgery and provide an up-to-date summary of its reported outcomes, safety, and complications.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7023973/

Record 843

pmcid: PMC7368203

pmid: 32695532

doi: 10.1155/2020/1928121

title: Association of Central Precocious Puberty with a Rare Presentation of Schimmelpenning-Feuerstein-Mims Syndrome in a Peruvian Girl.

authors

- De Los Santos-La Torre MA
- Del Águila-Villar CM
- Lu-de Lama LR
- Nuñez-Almache O
- Chávez-Tejada EM
- Espinoza-Robles OA
- Pinto-Ibárcena PM
- Calagua-Quispe MR

journal: Case reports in endocrinology

journalAbbr: Case Rep Endocrinol

year: 2020

pubdate: 2020

volume: 2020

issue:

pages: 1928121

abstract: Schimmelpenning-Feuerstein-Mims syndrome (SFM), an epidermal nevus syndrome characterized by skin lesions, has an estimated incidence of 1 per 10 000 live births. Nevus sebaceous, the most common cutaneous lesion, and verrucous nevus, the less frequent lesion, are coupled with a wide range of extracutaneous manifestations. As part of these manifestations, rarely, central precocious puberty can arise. We report the case of a 1-year-5-month-old girl who presented to the Endocrinology and Metabolism Department with breast enlargement that began at one year of age, growth of pubic and axillary hair three months later, and vaginal bleeding that occurred five months later. During clinical examination, melanocytic nevi, with a diameter ranging from 3 to 5 mm, were noted on the face. Verrucous nevi of variable size with a tendency for coalescence following the lines of Blaschko and melanocytic nevi with a diameter ranging from 3 to 6 mm were observed on the right hemibody and on the left hemibody, respectively. Right asymmetry of the lower extremities was observed. Laboratory findings showed a significant increase in the levels of follicle-stimulating hormone (FSH) and luteinizing hormone (LH) after the gonadotropin-releasing hormone (GnRH) stimulation test; additionally, imaging demonstrated advanced bone age and pubertal changes in the internal genitals. Analyses of the H-RAS, K-RAS, and N-RAS genes in the blood and in the skin were performed, and a missense mutation in exon 2 of the gene, H-RAS c37G > C (p.G13R), was detected in the latter. Treatment with triptorelin, a GnRH analog, was initiated, and it gave good clinical response. Epidermal nevus syndrome has a wide and variable systemic involvement. Thus, it is important to consider the development of precocious puberty for a prompt diagnosis and to strategize a multidisciplinary approach from the beginning.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7368203/

Record 844

pmcid: PMC7519915

pmid: 32449041

doi: 10.1007/s00296-020-04605-5

title: Autoimmune syndrome induced by adjuvants after breast enhancement with polyacrylamide hydrogel: a study in Poland.

authors

- Woźniak-Roszkowska E
- Maślińska M
- Gierej P
- Noszczyk B

journal: Rheumatology international

journalAbbr: Rheumatol Int

year: 2020

pubdate: 2020 Nov

volume: 40

issue: 11

pages: 1851-1856

abstract: Autoimmune syndrome induced by adjuvants (ASIA) is the spectrum of diseases in which the

substances considered inert to the body induce autoimmune reactions and inflammation. Some of the biomaterials recently used in plastic surgery, such as silicone or polyacrylamide hydrogel (PAAG) seem to trigger clinical features of ASIA. The aim of this study was to assess the incidence of these features within a group of women after breast augmentation with PAAG. As many as 30 consecutive patients (26-59 years, mean age 39.5) referred to the Clinic of Plastic Surgery after breast enhancement with PAAG were examined. The validated criteria of ASIA syndrome were employed. Descriptive statistics were chosen based on the distribution of variables. The research was approved by the Bioethical Committee of the Centre of Postgraduate Medical Education in Warsaw, Poland. Within the studied group, 50% of patients (n = 15) fulfilled ASIA diagnostic criteria. Apart from local complications, we encountered various general symptoms, among which fever (n = 13, 43.3%), tingling and numbness of upper extremities (n = 10, 33.3%) and chronic fatigue (n = 9, 30%) were the most common. These symptoms were present on an ambulatory visit, before qualification to the operation of hydrogel removal. All patients undergoing surgical PAAG removal (n = 8) declared alleviation or complete resolution of the symptoms. Polyacrylamide hydrogel breast filling, although limiting the invasiveness of the procedure in relation to silicone breast implants, also carries the risk of developing ASIA symptoms. The removal of PAAG may bring improvement in some cases.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7519915/

Record 845

pmcid: PMC7417367

pmid: 32849305

doi: 10.3389/fendo.2020.00522

title: Auxological and Endocrinological Features in Children With McCune Albright Syndrome: A Review.

authors

- Tufano M
- Ciofi D
- Amendolea A
- Stagi S

journal: Frontiers in endocrinology

journalAbbr: Front Endocrinol (Lausanne)

year: 2020

pubdate: 2020

volume: 11

issue:

pages: 522

abstract: McCune-Albright syndrome is a rare and challenging congenital sporadic disease involving the skin and skeletal and endocrine systems with a prevalence ranges from one in 100,000 to 1,000,000. In addition to the classical triad of fibrous dysplasia of bone, café au lait pigmented skin lesions and precocious puberty, other multiple endocrinological features, including hyperthyroidism, growth hormone excess, hypercortisolism, and hypophosphatemic rickets, have been reported. A brief review of the syndrome in children is here reported.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7417367/

Record 846

pmcid: PMC7251487

pmid: 32454451

doi: 10.1016/j.ijscr.2020.05.001

title: Axillary enlargement in gestational gigantomastia, an uncommon presentation of a rare disease. A case

report.

authors

- Athamnah MN
- Al-Share MS
- Hajjaj EM
- Athamneh LN
- Abuelaish OM

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2020

pubdate: 2020

volume: 71

issue:

pages: 155-158

abstract: BACKGROUND: Gestational gigantomastia presents in pregnancy as a rapid and excessive enlargement of the breast reducing the quality of life and causing psychological and physical impairment. Huge axillary enlargement due to Gestational gigantomastia is not mentioned in the literature before. Best treatment remains unclear as conservative and medical management are options beside the more aggressive surgical treatment. CASE PRESENTATION: This is a special case of Gestational gigantomastia presented by huge axillary and breast enlargement in a 34-year-old female. Treated successfully in a conservative manner, a near complete spontaneous resolution happened two months post-delivery. CONCLUSION: Axillary involvement is not mentioned in the limited literature discussing the rare disease of gestational gigantomastia. Not only breast but also axillary area can be involved and enlarge due to this disease. Conservative non-operative management was successful.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7251487/

Record 847

pmcid: PMC7585869

pmid: 33133326

doi: 10.1016/j.radcr.2020.10.015

title: Breast hamartoma: Unusual radiological presentation.

authors

- Mohamed A

journal: Radiology case reports

journalAbbr: Radiol Case Rep

year: 2020

pubdate: 2020 Dec

volume: 15

issue: 12

pages: 2714-2717

abstract: Breast hamartomas are uncommon slowly growing lesions. Imaging findings are specific and usually relay on common mammographic picture of breast within breast appearance. I present a case of 48 years female patient, with recent history of palpable right breast mass, the imaging findings are atypical for breast

hamartoma. The unusual presentation of breast hamartomas necessitates biopsy to exclude tumor. Incidence of malignancy is low, however has been reported before. Characteristic radiological imaging can help to minimize unnecessary surgery and morbidity.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7585869/

Record 848

pmcid: PMC7325414

pmid: 32617230

doi: 10.7759/cureus.8359

title: Breast Imaging Chameleon: Pseudoangiomatous Stromal Hyperplasia Presenting as Breast Malignancy.

authors

- Raza R
- Fatima K
- Tariq MU

journal: Cureus

journalAbbr: Cureus

year: 2020

pubdate: 2020 May 30

volume: 12

issue: 5

pages: e8359

abstract: Pseudoangiomatous stromal hyperplasia (PASH) is a benign mesenchymal proliferative lesion of the breast, often an incidental finding on breast biopsy specimens and rarely presents as a palpable lump. The case being reported is interesting as a lactating female presented with gross left breast enlargement due to a huge firm mass with skin thickening and palpable left axillary lymph nodes. A provisional diagnosis of left breast malignancy was made and the patient extensively worked up with ultrasound, CT scan, bone scan and core biopsy. The histopathology, however, revealed PASH of the breast. There was no invasive or in situ malignancy. The patient was successfully managed conservatively.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7325414/

Record 849

pmcid: PMC7552810

pmid: 33082901

doi: 10.1016/j.radcr.2020.09.033

title: Breast implant-associated anaplastic large cell lymphoma with contralateral invasive lobular carcinoma.

authors

- Johnson L
- Lowry K
- Scheel J
- Mau B
- Rockoff SJ

journal: Radiology case reports

journalAbbr: Radiol Case Rep

year: 2020

pubdate: 2020 Dec

volume: 15

issue: 12

pages: 2572-2576

abstract: Breast implant-associated anaplastic large cell lymphoma (BIA-ALCL) is a rare type of T-cell lymphoma that arises in the setting of textured breast implants. In this case report, a 69-year-old woman with a remote history of right-sided invasive lobular carcinoma status post right mastectomy and bilateral breast reconstruction presents with spontaneous right breast swelling and pain, suspicious for implant rupture. Diagnostic MRI revealed a peri-implant fluid collection in the right breast and focal nonmass enhancement in the left breast. The patient was ultimately diagnosed with right-sided BIA-ALCL and left-sided invasive lobular carcinoma. Although intravenous gadolinium contrast is not needed to assess implant integrity, it can be used to evaluate for malignancy when the patient is at an increased risk for developing breast cancer. In this case, the use of contrast revealed the rare instance of a synchronous contralateral invasive lobular carcinoma. Despite the rarity of BIA-ALCL with an estimated incidence of 1:30,000 in women with textured implants, it is essential that radiologists include this entity in the differential in the appropriate clinical setting as surgical resection is curative if performed before the disease has spread.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7552810/

Record 850

pmcid: PMC7275238

pmid: 32550034

doi: 10.1155/2020/2157485

title: Breast Implant-Associated Anaplastic Large Cell Lymphoma.

authors

- Moori PL
- Ibison F
- Jacob D
- Iddon J

journal: Case reports in pathology

journalAbbr: Case Rep Pathol

year: 2020

pubdate: 2020

volume: 2020

issue:

pages: 2157485

abstract: Breast implant-associated anaplastic large cell lymphoma (BIA-ALCL) is a rare, non-Hodgkin lymphoma which arises within the capsules of breast implants. These particular tumours have expression of CD30 and are negative for Anaplastic Lymphoma Kinase (ALK). Here, we report a case of BIA-ALCL in a 48-year-old woman post breast reconstruction. This case report is aimed at raising awareness and education on the significance of considering the development of BIA-ALCL in cases where cytology is negative and helping better understand this disease process.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7275238/

Record 851

pmcid: PMC7315302

pmid: 32114749

doi: 10.14366/usg.19070

title: Breast lesions during pregnancy and lactation: a pictorial essay.

authors

- Lee SE
- Bae YK

journal: Ultrasonography (Seoul, Korea)

journalAbbr: Ultrasonography

year: 2020

pubdate: 2020 Jul

volume: 39

issue: 3

pages: 298-310

abstract: A wide range of breast lesions can arise during pregnancy and lactation, from benign or Epub ahead of print inflammatory diseases to malignant tumors. Hormone-influenced physiological changes of the breast make the radiological evaluation of breast lesions challenging. Knowledge of the imaging features of normal physiological changes and common breast lesions during this period can help radiologists accurately diagnose and appropriately manage conditions. As such, this pictorial essay illustrates normal physiological changes related to pregnancy and lactation, as well as common benign and malignant breast lesions encountered during those periods.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7315302/

Record 852

pmcid: PMC7350395

pmid: 32694924

doi: 10.1177/1179547620933585

title: Case Report of Central Precocious Puberty in a Female: Role of Environmental Pollutants?

authors

- Helyaich A
- Elqadiry R
- El Fakiri K
- Rada N
- Draiss G
- Bouskraoui M

journal: Clinical medicine insights. Case reports

journalAbbr: Clin Med Insights Case Rep

year: 2020

pubdate: 2020

volume: 13

issue:

pages: 1179547620933585

abstract: A 5-year-old girl was brought to Department of Pediatric Endocrinology and Diabetes for premature breast development since 4 months. Her medical antecedents and family history were uneventful. From investigations she was diagnosed as a case of central precocious puberty. Identification of pesticides in farms surrounding their house indicates that this early stimulation of the hypothalamic-pituitary-gonadal axis was linked to the estrogen-like activity of endocrine-disrupting compounds.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7350395/

Record 853

pmcid: PMC7838954

pmid: 33325409

doi: 10.3233/PRM-200728

title: Central precocious puberty in spina bifida children: Guidelines for the care of people with spina bifida.

authors

- Almutlaq N
- O'Neil J
- Fuqua JS

journal: Journal of pediatric rehabilitation medicine

journalAbbr: J Pediatr Rehabil Med

year: 2020

pubdate: 2020

volume: 13

issue: 4

pages: 557-563

abstract: Children with spina bifida are at greater risk of developing central precocious puberty (CPP) compared to others. Therefore, early recognition and timely referral for further evaluation by a pediatric endocrinologist allows appropriate management that reduces the impact of CPP. This article discusses the diagnosis and management of CPP in children with spina bifida. This guideline was developed for SB Transition Healthcare Guidelines from the 2018 Spina Bifida Association's Fourth Edition of the Guidelines for the Care of People with Spina Bifida.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7838954/

Record 854

pmcid: PMC7282750

pmid: 32619773

doi: 10.1016/j.clinimag.2020.06.004

title: Chest-CT findings of COVID-19 in patients with pre-existing malignancies; a pictorial review.

authors

- Katal S
- Aghaghazvini L
- Gholamrezanezhad A

journal: Clinical imaging

journalAbbr: Clin Imaging

year: 2020

pubdate: 2020 Nov

volume: 67

issue:

pages: 121-129

abstract: As of April 17th, 2020, more than 2,190,010 COVID-19 cases with 147,010 deaths have been recorded worldwide. It has been suggested that a high mortality rate occurs in patients with severe disease and is associated with advanced age and underlying comorbidities, such as malignancies. To the best of our knowledge, no study has been conducted to evaluate chest CT features in patients with malignancy and concomitant COVID-19 infection. In fact, the imaging findings can be challenging and have not yet been fully understood in this setting. In this manuscript, we go over imaging findings in chest CT of patients with COVID-19 and known cancer. With the ongoing COVID-19 pandemic and exponentially increasing incidence throughout the world, in at-risk and vulnerable populations such as patients with known malignancies, infection with SARS-CoV-2 should be included in the differential considerations even with atypical image pictures. Detection of superimposed infection in patients with cancers who present with pulmonary infiltrations warrant correlation with clinical picture, contact history, and RT-PCR confirmatory testing.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7282750/

Record 855

pmcid: PMC7179388

pmid: 32337041

doi: 10.3892/mco.2020.2025

title: Clinicopathological features and maternal and foetal management of pregnancy-complicating Krukenberg tumours.

authors

- Zhang J
- Peng P
- Cao D
- Yang J
- Shen K

journal: Molecular and clinical oncology

journalAbbr: Mol Clin Oncol

year: 2020

pubdate: 2020 Jun

volume: 12

issue: 6

pages: 581-587

abstract: Krukenberg tumours are not uncommon, but pregnancy-complicating Krukenberg tumours are rare. To identify management strategies of pregnancies with Krukenberg tumours, the medical records of patients treated at Peking Union Medical College Hospital over the past 20 years were collected and analysed. Four patients were enrolled. The primary tumour sites were the stomach and colorectal region. Three patients presented with obvious and severe gastrointestinal symptoms at 11-18 weeks of gestation, and their symptoms gradually developed into physiological signs associated with malignancy. Three patients underwent termination of pregnancy via induced delivery and gestational hysterectomy. One patient underwent caesarean section at 31 weeks of gestation. Two patients underwent palliative surgery due to extensive lesions. The other two patients with metastatic lesions confined to the ovaries and liver, underwent satisfactory cytoreductive surgery and anti-tumour treatments postoperatively. Pregnant women who exhibit new gastrointestinal symptoms or

aggravation of previous gastrointestinal symptoms after the end of the first trimester should be carefully examined for digestive system diseases. If ovarian metastasis is highly suspected, the pregnancy should be terminated as early as possible.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7179388/

Record 856

pmcid: PMC7804523

pmid: 33447570

doi: 10.21037/gs-20-448

title: Complications after breast augmentation with hyaluronic acid: a case report.

authors

- Trignano E
- Baccari M
- Pili N
- Serra PL
- Rubino C

journal: Gland surgery

journalAbbr: Gland Surg

year: 2020

pubdate: 2020 Dec

volume: 9

issue: 6

pages: 2193-2197

abstract: Breast augmentation is a very requested procedure in aesthetic surgery. Over time, various techniques of breast augmentation have been described, both through the use of alloplastic materials and autologous tissues: implants, fat grafting, flap surgery, and injectable fillers. According to the international current trend to perform minimally invasive procedure, injectable fillers have progressively increased their uses and notoriety in aesthetic medicine. Among the various available products, hyaluronic acid is likely to be the most popular synthetic filler all over the world. Nowadays many brands are on the market and this paper focuses on Macrolane which is a NASHA-based (stabilized hyaluronic acid of non-animal origin) filler. However, although highly biocompatible, as any material, foreign to our body, also hyaluronic acid fillers undergo a process of degradation and disposal. In our work, we report the first case in literature of migration of Macrolane in an axillary lymph node in a 45-year-old woman three years after injection for breast augmentation and its related management. The aim of the paper is to discuss and underline further complications and their treatment besides the ones already described in literature in order to provide a deeper knowledge concerning the use and the side effects of injectable fillers in aesthetic medicine.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7804523/

Record 857

pmcid: PMC7464587

pmid: 32905168

doi: 10.1093/jscr/rjaa276

title: Concentric mastopexy for pubertal gynaecomastia in an African child.

authors

- Michael AI

- Olawoye OA
- Oluwatosin OM

journal: Journal of surgical case reports

journalAbbr: J Surg Case Rep

year: 2020

pubdate: 2020 Sep

volume: 2020

issue: 9

pages: rjaa276

abstract: Gynaecomastia refers to the enlargement of the male breast. It can be psychologically disturbing for children and adolescents as well as adults. While in children watchful waiting is advised as the gynaecomastia could resolve, surgical intervention becomes necessary where there is significant psychosocial disturbance. We present a case of an African child whose quality of life improved after correcting the gynaecomastia. We highlight the concentric mastopexy method in the surgical management of gynaecomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7464587/

Record 858

pmcid: PMC8188909

pmid: 34158724

doi: 10.4103/ipj.ipj_62_19

title: Delusion of pregnancy for 21 years in an elderly woman: A case report and review of literature.

authors

- Dua D
- Grover S

journal: Industrial psychiatry journal

journalAbbr: Ind Psychiatry J

year: 2020

pubdate: 2020 Jul-Dec

volume: 29

issue: 2

pages: 342-344

abstract: Delusion of pregnancy (DOP) is a rare phenomenon especially in the elderly, with only 22 cases being reported. In this report, we present a case of a 74-year-old female with DOP and depressive disorder. In this case, a diagnosis of persistent delusional disorder was considered as the DOP persisted even after resolution of depressive symptoms with the use of antidepressants, antipsychotics, and electroconvulsive therapy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8188909/

Record 859

pmcid: PMC7225459

pmid: 32420252

doi: 10.21037/gs.2020.03.30

title: Differential diagnosis of plasma cell mastitis and invasive ductal carcinoma using multiparametric MRI.

authors

- Chen R
- Hu B
- Zhang Y
- Liu C
- Zhao L
- Jiang Y
- Xu Y

journal: Gland surgery

journalAbbr: Gland Surg

year: 2020

pubdate: 2020 Apr

volume: 9

issue: 2

pages: 278-290

abstract: BACKGROUND: Evaluate the potential of multiparametric magnetic resonance imaging (MRI) for the differential diagnosis of plasma cell mastitis (PCM) and invasive ductal carcinoma (IDC). METHODS: A total of 465 female patients, including 197 PCM (42.4%) and 268 IDC (57.6%), were examined using breast MRI scanning using routine sequences, dynamic contrast-enhanced MRI (DCE-MRI), diffusion-weighted imaging (DWI) and MR spectroscopy (MRS). The MRI features of PCM and IDC lesions were analyzed and compared to the histological results. RESULTS: Compared to IDC, the PCM lesions were more frequent in the subareolar regions, hyperintense on T2WI (P<0.01) and showed an initial signal increase ≤90%, a persistent and plateau pattern of time-intensity curves, non-mass enhancement, multiple rim enhancements, central hyperintensity on DWI, a higher ADC value, and total choline (tCho) peak negative and tCho peak integral <29.95 AU (P<0.01). The following breast-associated findings were also observed frequently in PCM: Ipsilateral breast enlargement, nipple retraction, skin thickening, peripheral edema and axillary lymphadenopathy. However, no significant difference was observed between the two groups for the shape, border and adjacent vessel signs of the lesion. CONCLUSIONS: Some of the MRI features of PCM and IDC lesions were different. An integrated analysis of these multiparametric MRI features can thus assist in the differential diagnosis of PCM and IDC lesions.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7225459/

Record 860

pmcid: PMC7521058

pmid: 32967508

doi: 10.1177/0300060520954694

title: Dual effects of a gonadotropin-releasing hormone agonist on an adolescent girl with pelvic congestion syndrome and precocious puberty: a case report.

authors

- Yu JH
- Fang HH
- Liu SY
- Chang WC
- Liu CC
- Lin CM

journal: The Journal of international medical research

journalAbbr: J Int Med Res

year: 2020

pubdate: 2020 Sep

volume: 48

issue: 9

pages: 300060520954694

abstract: Pelvic congestion syndrome (PCS) typically causes chronic non-cyclical abdominal pain with a considerable negative effect on the quality of life of women. However, pediatric cases with PCS are limited and non-invasive therapy for adolescent patients has not been reported. We report here a 13-year-old girl who presented with intermittent abdominal pain since the age of 2 years and her symptoms further deteriorated after breast development at 6 years and 9 months old. PCS and coexistent idiopathic central precocious puberty were finally diagnosed on the basis of tortuous ovarian and pelvic veins, and a pubertal response to a gonadotropin-releasing hormone (GnRH) test without hypothalamic-pituitary lesions. After treatment with the GnRH agonist, the pain score was greatly reduced and there was increased prediction of adult height. This case highlights the occurrence of PCS in adolescents and also indicates the role of non-invasive GnRH agonists in young patients with PCS before surgical intervention.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7521058/

Record 861

pmcid: PMC7643790

pmid: 33136923

doi: 10.1097/JCP.0000000000001297

title: Effect of Brexpiprazole on Prolactin and Sexual Functioning: An Analysis of Short- and Long-Term Study Data in Major Depressive Disorder.

authors

- Clayton AH
- Ivkovic J
- Chen D
- George V
- Hobart M

journal: Journal of clinical psychopharmacology

journalAbbr: J Clin Psychopharmacol

year: 2020

pubdate: 2020 Nov/Dec

volume: 40

issue: 6

pages: 560-567

abstract: PURPOSE/BACKGROUND: Evidence supports use of adjunctive atypical antipsychotics in major depressive disorder (MDD). Impaired sexual functioning is common in MDD and may be worsened by antipsychotic adverse effects. We evaluated the effect of brexpiprazole on prolactin and sexual functioning in patients with MDD. METHODS/PROCEDURES: In short-term studies, patients received adjunctive brexpiprazole 1, 2, or 3 mg or placebo. The long-term study was a flexible-dose (0.5-3 mg/d) open-label extension (OLE). Change from baseline and shifts in prolactin status and prolactin-related treatment-emergent adverse events (TEAEs) were assessed. Sexual functioning was assessed by the Massachusetts General Hospital Sexual Functioning Questionnaire. FINDINGS/RESULTS: Median changes in prolactin levels from

baseline to week 6 in short-term studies were as follows: brexpiprazole, 5.99 ng/mL (females) and 1.61 ng/mL (males); placebo, -0.15 ng/mL (females) and -0.08 ng/mL (males).Median changes from baseline to week 52 in the OLE were as follows: 0.27 ng/mL (females) and 0.27 ng/mL (males). Prolactin levels in patients with baseline prolactin greater than 1× upper limit of normal values tended to decrease over time.The proportion of brexpiprazole-treated patients with greater than 3× upper limit of normal postbaseline prolactin values in short-term studies for both sexes was low (0%-0.3%) and did not differ from placebo: OLE, 0.5% (females) and 0.8% (males).In short-term studies, the incidence of prolactin-related TEAEs was 3.1% for brexpiprazole and 0.7% for placebo (OLE, 3.1%). There were overall numerical improvements from baseline in sexual functioning for females and males after short- and long-term brexpiprazole treatment, with statistically significant improvements for brexpiprazole versus placebo in females on the items 'interest in sex' (-0.19; 95% confidence interval [CI], -0.33 to -0.05; P = 0.0074), 'sexually aroused' (-0.17; 95% CI, -0.30 to -0.03; P = 0.0154), and 'overall sexual satisfaction' (-0.16; 95% CI, -0.30 to -0.03; P = 0.0184). IMPLICATIONS/CONCLUSIONS: There were small changes in prolactin levels, low proportions of patients with postbaseline elevated prolactin values, low incidences of prolactin-related TEAEs, and modest improvements in sexual functioning with adjunctive brexpiprazole in MDD.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7643790/

Record 862

pmcid: PMC7225336

pmid: 32457631

doi: 10.3389/fphar.2020.00658

title: Efficacy and Side Effects of Drugs Commonly Used for the Treatment of Lower Urinary Tract Symptoms Associated With Benign Prostatic Hyperplasia.

authors

- Yu ZJ
- Yan HL
- Xu FH
- Chao HC
- Deng LH
- Xu XD
- Huang JB
- Zeng T

journal: Frontiers in pharmacology

journalAbbr: Front Pharmacol

year: 2020

pubdate: 2020

volume: 11

issue:

pages: 658

abstract: Benign prostatic hyperplasia (BPH) is the most common benign disease of the prostate gland and is caused by benign hyperplasia of the smooth muscle cells and stromal cells in this important gland. BPH is also the most common disease underlying lower urinary tract symptoms (LUTS). The incidence of BPH increases with age and affects more than half of all men 50 years or older. BPH mainly exerts effects on urinary function and can seriously reduce a patient's quality of life. At present, treatment for BPH aims primarily to improve the quality of life and reduce the risk of BPH-related complications. Pharmacological therapy is recommended for moderate-to-severe cases of LUTS that are suggestive of BPH. A range of drugs is currently available to treat this condition, including α1-adrenoceptor antagonists, 5α-reductase inhibitors (5-ARIs), phosphodiesterase type

5 inhibitors (PDE5Is), muscarinic receptor antagonists (MRAs), β3-adrenoceptor agonists, and plant extracts. Of these, the most commonly used drugs in the clinic are α1-adrenoceptor antagonists, 5-ARIs, and combination therapy. However, these drugs exert their effects via various mechanisms and are associated with adverse reactions. The purpose of this review is to provide current comprehensive perspectives on the mechanisms of action, efficacy, and adverse reactions associated with the drugs most commonly used for the treatment of BPH.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7225336/

Record 863

pmcid: PMC7260267

pmid: 31858387

doi: 10.1007/s11673-019-09959-5

title: Elective Impairment Minus Elective Disability: The Social Model of Disability and Body Integrity Identity Disorder.

authors

- Gibson RB

journal: Journal of bioethical inquiry

journalAbbr: J Bioeth Inq

year: 2020

pubdate: 2020 Mar

volume: 17

issue: 1

pages: 145-155

abstract: Individuals with body integrity identity disorder (BIID) seek to address a non-delusional incongruity between their body image and their physical embodiment, sometimes via the surgical amputation of healthy body parts. Opponents to the provision of therapeutic healthy-limb amputation in cases of BIID make appeals to the envisioned harms that such an intervention would cause, harms such as the creation of a lifelong physical disability where none existed before. However, this concept of harm is often based on a normative biomedical model of health and disability, a model which conflates amputation with impairment, and impairment with a disability. This article challenges the prima facie harms assumed to be inherent in limb amputation and argues in favour of a potential treatment option for those with BIID. To do this, it employs the social model of disability as a means to separate the concept of impairment and disability and thereby separate the acute and chronic harms of the practice of therapeutic healthy-limb amputation. It will then argue that provided sufficient measures are put in place to ensure that those with atypical bodily constructions are not disadvantaged, the chronic harms of elective amputation would cease to be.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7260267/

Record 864

pmcid: PMC7735009

pmid: 33344347

doi: 10.4103/idoj.IDOJ_287_20

title: Endocrinological Testicular Dysfunction in Patients with Lepromatous Leprosy and the Impact of Disease on Patient's Quality of Life.

authors

- Mohta A
- Agrawal A

- Sharma P
- Singh A
- Garg S
- Kushwaha RK
- Jain SK

journal: Indian dermatology online journal

journalAbbr: Indian Dermatol Online J

year: 2020

pubdate: 2020 Nov-Dec

volume: 11

issue: 6

pages: 959-964

abstract: INTRODUCTION: Leprosy or Hansen's disease poses a drastic impact on the quality of life in affected patients even after successful completion of treatment. The involvement of the endocrine system in leprosy is usually insidious, silent, and under-reported, especially the testicular dysfunction. AIMS AND OBJECTIVES: The present study was aimed at evaluating the abnormalities of the primary testicular hormone testosterone and the gonadotrophins LH and FSH in male patients affected with lepromatous leprosy and assessing the impact of the disease on quality of life (QOL). MATERIALS AND METHODS: The study included 43 married male patients diagnosed with lepromatous leprosy. Careful history taking and examination for symptoms of testicular dysfunction were done. Serum concentrations of total testosterone, FSH, and LH were noted. The QOL was evaluated using the WHO Quality of Life-BREF (WHOQoL-BREF). RESULTS: The most common clinical manifestation of testicular dysfunction was reduced or loss of libido reported in 12 (27.9%) patients followed by gynaecomastia in 7 (16.3%). Ultrasonographic (USG) analysis revealed reduced testicular volume in 31 (72.1%) patients, and average testicular volume was 11.9 ± 4.9 mL each. Seventeen (39.5%) patients had low serum testosterone levels, 9 (20.9%) had high serum FSH level, and 11 (25.6%) high LH levels. There was a significant negative correlation between testosterone level and FSH as well as LH. There was also a significantly positive correlation between testicular volume and testosterone level. Symptomatic patients with gynaecomastia/gynaecothelia had higher hormonal derangement than those who had other symptoms. On QOL, most patients scored lowest on the domain of "social relationships" (including sexual wellbeing) followed by "psychological health". CONCLUSION: We found a high rate of USG diagnosed testicular atrophy in lepromatous leprosy patients. Therefore, every leprosy patient should be thoroughly examined clinically to rule out features of testicular dysfunction. Testicular function tests should be routinely carried out in all leprosy patients to arrive at an early diagnosis. Leprosy is found to affect all domains of a patient's quality of life.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7735009/

Record 865

pmcid: PMC7581442

pmid: 32639099

doi: 10.1002/sctm.20-0081

title: Ex vivo-expanded autologous adipose tissue-derived stromal cells ensure enhanced fat graft retention in breast augmentation: A randomized controlled clinical trial.

authors

- Kølle ST
- Duscher D
- Taudorf M
- Fischer-Nielsen A
- Svalgaard JD

- Munthe-Fog L
- Jønsson B
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- Mamsen FP
- Katz AJ

journal: Stem cells translational medicine

journalAbbr: Stem Cells Transl Med

year: 2020

pubdate: 2020 Nov

volume: 9

issue: 11

pages: 1277-1286

abstract: Autologous fat grafting and implant surgery are used for volume restoration in plastic surgery. With the aim of producing a treatment superior to current solutions, we report a randomized, controlled, data assessor-blinded clinical trial comparing fat grafts enriched with ex vivo-expanded autologous adipose-derived stromal cells (ASCs) to nonenriched fat grafts in breast augmentation. The intervention group received ASC-enriched fat grafts (≥20 × 10 6 viable ex vivo-expanded ASCs per milliliter fat), and the control group received conventional nonenriched fat grafts. Volume retention was measured by magnetic resonance imaging, and clinical photographs were taken simultaneously for outcome evaluation. ASC-enriched fat grafts had significantly higher retention rates (mean = 80.2%) compared with conventional fat grafts (mean = 45.1%). Clinical photos showed statistically significant superior results in the intervention group, assessed by independent clinical experts. These results improve the prospects for using culture-expanded ASCs in both reconstructive and cosmetic volume restoration and make the procedure an attractive alternative to conventional fat grafting and implants. This study is registered at www.ClinicalTrials.gov, number H-16046960.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7581442/

Record 866

pmcid: PMC7318446

pmid: 32586344

doi: 10.1186/s13063-020-04432-w

title: FASCE, the benefit of spironolactone for treating acne in women: study protocol for a randomized double-blind trial.

authors

- Poinas A
- Lemoigne M
- Le Naour S
- Nguyen JM
- Schirr-Bonnans S
- Riche VP
- Vrignaud F
- Machet L
- Claudel JP
- Leccia MT
- Hainaut E
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- Dert C
- Boisrobert A
- Flet L
- Chiffoleau A
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- Khammari A
- Dréno B

journal: Trials

journalAbbr: Trials

year: 2020

pubdate: 2020 Jun 25

volume: 21

issue: 1

pages: 571

abstract: BACKGROUND: Acne vulgaris has increased in women over the past 10 years; it currently affects 20-30% of women. The physiopathology of adult female acne is distinguished from that of teenagers essentially by two factors: hormonal and inflammatory. On a therapeutic plan, the four types of systemic treatment approved for female acne include cyclines (leading to bacterial resistance); zinc salts (less effective than cyclines); and antiandrogens (risks of phlebitis). The last alternative is represented by isotretinoin, but its use in women of childbearing potential is discouraged because of the teratogen risks. In this context, spironolactone could represent an interesting alternative. It blocks the 5-alpha-reductase receptors at the sebaceous gland and inhibits luteinizing hormone (LH) production at the pituitary level. It has no isotretinoin constraints and does not lead to bacterial resistance. Currently, very few studies have been performed in a limited number of patients: the studies showed that at low doses (lower than 200 mg/day), spironolactone can be effective against acne. In that context, it is clearly of interest to perform the first double-blind randomized study of spironolactone versus cyclines, which remains the moderate acne reference treatment, and to demonstrate the superiority of spironolactone's efficacy in order to establish it as an alternative to cyclines. METHODS: Two hundred female patients will be included. They must have acne vulgaris with at least 10 inflammatory lesions and no more than 3 nodules. After randomization, the patients will be treated by spironolactone or doxycycline for 3 months and after placebo. The study will be blind for the first 6 months and open for the last 6 months. DISCUSSION: The treatment frequently used in female acne is systemic antibiotics with many courses, as it is a chronic inflammatory disease. In the context of the recent World Health Organisation (WHO) revelation about the serious, worldwide threat to public health of antibiotic resistance, this trial could give the physician another alternative in the treatment of adult female acne instead of using isotretinoin, which is more complex to manage. TRIAL REGISTRATION: ClinicalTrials.gov: NCT03334682. Registered on 7 November 2017.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7318446/

Record 867

pmcid: PMC7653081

pmid: 33166811

doi: 10.1016/j.ijscr.2020.10.080

title: Gestational gigantomastia on a Saudi woman: A case report on surgical removal and reconstruction and management of complications, KFSH&RC.

authors

- Mahabbat N
- Abdulla A
- Alsufayan F

- Alharbi A
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- Hashem F

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2020

pubdate: 2020

volume: 77

issue:

pages: 157-160

abstract: INTRODUCTION: Gestational gigantomastia (GG) is a rare condition manifesting as a fast and excessive growth of the breasts in pregnant women. Its etiology is still unclear, with theories ranging from hormonal imbalances, unregulated immune response, to hypersensitivity. Medical interventions are mainly surgical in nature, though some pharmacological medications are of debatable efficacy. CASE PRESENTATION: A 33-year old Saudi gravida 3 para 2 presents continuous breast enlargement since the start of her pregnancy. She complains of skin ulcerations and discharge which was initially treated conservatively with topical antibiotics. Days after she came back with worsening GG symptoms, and was admitted for bilateral skin sparing mastectomy and reconstruction, and successfully recovered. The patient came back with problems concerning the surgical implant and wound infection. Emergency operation was performed for implant removal and wound treatment. Labor induction was performed by the OB-GYN on her 39 week. The patient opted for autogenous reconstruction by bilateral latissimus dorsi flap months after delivery. After treatment of minor surgical complications, the patient successfully recovered. DISCUSSION: Surgery is one of the most effective interventions for GG. Total mastectomy is preferred due to lesser risk of recurrence in subsequent pregnancies. Reduction mammoplasty offers the breastfeeding option if conducted before the delivery, but poses higher risk of recurrence in future pregnancies. CONCLUSION: The patient's gestational gigantomastia condition was complicated by several ulcerations and infections. Surgery was conducted alongside antibiotic treatment. This report also highlights the importance of follow ups in managing complications.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7653081/

Record 868

pmcid: PMC7588656

pmid: 33134461

doi: 10.1016/j.jdcr.2020.07.016

title: Gestational gigantomastia: A case report and brief review of the literature.

authors

- Fletcher MB
- Corsini LM
- Meyer MD
- Osswald SS

journal: JAAD case reports

journalAbbr: JAAD Case Rep

year: 2020

pubdate: 2020 Nov

volume: 6

issue: 11

pages: 1159-1161

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7588656/

Record 869

pmcid: PMC7186636

pmid: 32355841

doi: 10.21037/atm.2020.02.41

title: Gynecological cancer among adolescents and young adults (AYA).

authors

- Triarico S
- Capozza MA
- Mastrangelo S
- Attinà G
- Maurizi P
- Ruggiero A

journal: Annals of translational medicine

journalAbbr: Ann Transl Med

year: 2020

pubdate: 2020 Mar

volume: 8

issue: 6

pages: 397

abstract: Adolescents and young adults (AYA) patients with cancer show specific biological, sociodemographic and behavioral features, with lower survival rates than younger group. Gynecologic malignancies that occur among AYA requires a multidisciplinary management and a tailored model of care, in order to enhance the early diagnosis, the adherence to the treatment, the enrollment in clinical trials, the rate of survival and the quality of life (QoL). In this article, we review the main gynecological tumors that may occur in AYA, with a focus on the clinical signs at the diagnosis and the modality of treatment. In addition, we proposed a model of multidisciplinary and personalized care for AYA with gynecological tumors, which can help the clinicians to manage the specific gynecologic concerns, such as ovarian failure, contraception, fertility, late psychosocial effects.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7186636/

Record 870

pmcid: PMC7324050

pmid: 32480405

doi: 10.1210/clinem/dgaa331

title: Heterozygous Deletions in MKRN3 Cause Central Precocious Puberty Without Prader-Willi Syndrome.

authors

- Meader BN
- Albano A

- Sekizkardes H
- Delaney A

journal: The Journal of clinical endocrinology and metabolism

journalAbbr: J Clin Endocrinol Metab

year: 2020

pubdate: 2020 Aug 1

volume: 105

issue: 8

pages: 2732-9

abstract: CONTEXT: Loss-of-function mutations in the imprinted genes MKRN3 and DLK1 cause central precocious puberty (CPP) but whole gene deletions have not been reported. Larger deletions of the chromosome 15q11-13 imprinted locus, including MKRN3, cause Prader-Willi syndrome (PWS). CPP has been reported in PWS but is not common, and the role of MKRN3 in PWS has not been fully elucidated. OBJECTIVE: To identify copy number variants in puberty-related, imprinted genes to determine their role in CPP. METHODS: Probands with idiopathic CPP had chromosomal microarray (CMA) and targeted deletion/duplication testing for MKRN3 and DLK1. RESULTS: Sixteen female probands without MKRN3 or DLK1 variants identified by Sanger sequencing were studied. Whole gene deletions of MKRN3 were identified in 2 subjects (13%): a complete deletion of MKRN3 in Patient A (pubertal onset at 7 years) and a larger deletion involving MAGEL2, MKRN3, and NDN in Patient B (pubertal onset 5.5 years). Both were paternally inherited. Patient B had no typical features of PWS, other than obesity, which was also present in her unaffected family. CONCLUSIONS: We identified 2 cases of whole gene deletions of MKRN3 causing isolated CPP without PWS. This is the first report of complete deletions of MKRN3 in patients with CPP, emphasizing the importance of including copy number variant analysis for MKRN3 mutation testing when a genetic diagnosis is suspected. We speculate that there is a critical region of the PWS locus beyond MKRN3, MAGEL2, and NDN that is responsible for the PWS phenotype.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7324050/

Record 871

pmcid: PMC7446157

pmid: 32838791

doi: 10.1186/s12916-020-01690-y

title: Hyperthyroidism is associated with breast cancer risk and mammographic and genetic risk predictors.

authors

- Yang H
- Holowko N
- Grassmann F
- Eriksson M
- Hall P
- Czene K

journal: BMC medicine

journalAbbr: BMC Med

year: 2020

pubdate: 2020 Aug 25

volume: 18

issue: 1

pages: 225

abstract: BACKGROUND: Despite the biological link between thyroid hormones and breast cancer cell proliferation shown in experimental studies, little is known about the association between hyperthyroidism and breast cancer, as well as its association with the most common mammographic and genetic risk predictors for breast cancer. METHODS: This study estimates the incidence rate ratios (IRRs) of breast cancer among women diagnosed with hyperthyroidism, compared to those who are not, using two cohorts: a Swedish national cohort of the general female population (n = 3,793,492, 2002-2011) and the Karolinska Mammography Project for Risk Prediction of Breast Cancer (KARMA, n = 69,598, 2002-2017). We used logistic regression to estimate the odds ratios (ORs) of hyperthyroidism according to the mammographic and genetic risk predictors for breast cancer. RESULTS: An increased risk of breast cancer was observed in patients in the national cohort with hyperthyroidism (IRR = 1.23, 95% CI = 1.12-1.36), particularly for toxic nodular goiter (IRR = 1.38, 95% CI = 1.16-1.63). Hyperthyroidism was associated with higher body mass index, early age at first birth, and lower breastfeeding duration. Higher mammographic density was observed in women with toxic nodular goiter, compared to women without hyperthyroidism. Additionally, among genotyped women without breast cancer in the KARMA cohort (N = 11,991), hyperthyroidism was associated with a high polygenic risk score (PRS) for breast cancer overall (OR = 1.98, 95% CI = 1.09-3.60) and for estrogen receptor-positive specific PRS (OR = 1.90, 95% CI = 1.04-3.43). CONCLUSION: Hyperthyroidism is associated with an increased risk of breast cancer, particularly for patients with toxic nodular goiter. The association could be explained by higher mammographic density among these women, as well as pleiotropic genetic variants determining shared hormonal/endocrine factors leading to the pathology of both diseases.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7446157/

Record 872

pmcid: PMC7775963

pmid: 33162384

doi: 10.1016/j.ijscr.2020.09.151

title: Idiopathic gigantomastia with Pseudoangiomatous stromal hyperplasia: A case report.

authors

- Vashistha A
- Rundla M
- Khan F
- Om P

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2020

pubdate: 2020

volume: 77

issue:

pages: 915-919

abstract: BACKGROUND: Gigantomastia is a rare condition characterized by excessive diffuse enlargement of both breasts and can be physically and psychosocially disabling for the patient. Despite an extensive search, the etiology remains poorly understood with most common being pubertal and gestational gigantomastia, with incidence of Idiopathic gigantomastia associated with bilateral PASH being extremely rare. METHODS: A 37 year old lady with bilateral gigantomastia and severe back pain with a normal radiological, hormonal and histopathological evaluation underwent reduction mammoplasty with objective of weight and volume reduction of the breasts along with aesthetic enhancement. RESULTS: The excised specimen weighed 3.5 and 5 kg respectively in left and right breast with uneventful post operative period and symptomatic relief to the patient. The histopathology was suggestive of macromastia with pseudoangiomatous stromal hyperplasia like areas with

focally PR positive status on IHC. CONCLUSION: Most commonly etiological factor for gigantomastia is endogenous hormone stimulation. While idiopathic gigantomastia is rare those associated with PASH are still rarer with around 13 cases reported in the literature till date. PASH is a beingn mesenchymal proliferative lesion of the breast, mostly found in premenopausal women and rarely manifests clinically. Reduction mammoplasty can make a significant improvement in life of such young patients with explained risk of probability of recurrence. Among the various techniques available inverted T scar pattern with superiomedical pedicle are preferred as the learning curve is shorter, have greater versatility, and is reproducible with consistent results.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7775963/

Record 873

pmcid: PMC7028902

pmid: 32072386

doi: 10.1186/s13244-019-0834-3

title: Imaging findings and classification of the common and uncommon male breast diseases.

authors

- Önder Ö
- Azizova A
- Durhan G
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journal: Insights into imaging

journalAbbr: Insights Imaging

year: 2020

pubdate: 2020 Feb 18

volume: 11

issue: 1

pages: 27

abstract: Male breast hosts various pathological conditions just like "female breast." However, histo-anatomical diversities with female breast lead to many differences regarding the frequency and presentation of diseases, the radiologic appearance of lesions, the diagnostic algorithm, and malignity features.Radiological modalities may play an important role in evaluating male breast lesions. Although some imaging findings are non-specific, having knowledge of certain imaging characteristics and radiologic patterns is the key to reduce the number of differential diagnoses or to reach an accurate diagnosis.Male breast imaging is mostly based on physical examination and is required for the complaints of palpable mass, breast enlargement, tenderness, nipple discharge, and nipple-skin changes. The majority of the male breast lumps are benign and the most common reason is gynecomastia. Although it is difficult to exclude malignancy in some cases, gynecomastia often has distinguishable imaging features. Pseudogynecomastia is another differential diagnosis that may be confused with gynecomastia. The distinction is important for the treatment plan.Apart from gynecomastia, other male breast lesions form a highly heterogeneous group and can be classified based on "Tissue origin," "Histopathological type and behavior," and "Radiologic features" for both simplification and comprehensive understanding.This article mainly focuses on emphasizing the results of basic histo-anatomical differences of male and female breasts, classifying male breast lesions, covering the spectrum of male breast diseases, and assisting radiologists in recognizing the imaging findings, in interpreting them through a holistic approach, in making a differential diagnosis, and in being a part of proper patient management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7028902/

Record 874

pmcid: PMC7278101

pmid: 32456508

doi: 10.1177/1557988320908102

title: Incidental Gynecomastia on Thoracic Computed Tomography in Clinical Practice: Characteristics, Radiologic Features, and Correlation With Possible Causes in South Korean Men.

authors

- Kim MS
- Kim JH
- Lee KH
- Suh YJ

journal: American journal of men's health

journalAbbr: Am J Mens Health

year: 2020

pubdate: 2020 May-Jun

volume: 14

issue: 3

pages: 1557988320908102

abstract: Gynecomastia is a common incidental finding on thoracic computed tomography (CT). This study aimed to retrospectively determine the prevalence, imaging characteristics, and possible causes of incidental gynecomastia on thoracic CT. Records of male patients who underwent thoracic CT in 2015 were reviewed. The size and morphologic types (nodular, dendritic, and diffuse) were recorded for patients with breast glandular tissue larger than 1 cm, and the cutoff value of gynecomastia was defined as 2 cm. Additionally, the possible causes of gynecomastia obtained by reviewing patients' charts were recorded. CT-depicted gynecomastia was identified in 12.7% (650 of 5,501) of patients. The median size of the breast glandular tissue was 2.5 cm (interquartile range 2.2-3.1), and 36.8% of patients (239 of 650) had unilateral gynecomastia. The age distribution provided a bimodal pattern with two peaks in the age groups from 20 to 29 years old and greater than 70 years old. Chronic liver disease (CLD; $p < .001$), all stages of chronic kidney disease (CKD; $p < .001$), and medications ($p = .002$) were significantly associated with gynecomastia. Gynecomastia did not correlate with body mass index ($p = .962$). The size of breast glandular tissue was identified to be correlated with the morphologic type of breast tissue and the severity of CLD or CKD. The prevalence of incidental gynecomastia seen on thoracic CT was 12.7%. CT-depicted gynecomastia is not associated with obesity but with CLD, CKD, and medications. When gynecomastia is detected on CT, further evaluations and management might be required for patients with a treatable cause.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7278101/

Record 875

pmcid: PMC7157970

pmid: 32290743

doi: 10.1177/0300060520918427

title: Individualization of computed tomography protocols for suspected pulmonary embolism: a national investigation of routines.

authors

- Dymbe B
- Mæland EV

- Styve JR
- Rusandu A

journal: The Journal of international medical research

journalAbbr: J Int Med Res

year: 2020

pubdate: 2020 Apr

volume: 48

issue: 4

pages: 300060520918427

abstract: OBJECTIVE: Given the extensive use of computed tomography (CT) in radiation-sensitive patients such as pregnant and pediatric patients, and considering the importance of tailoring CT protocols to patient characteristics for both the radiation dose and image quality, this study was performed to investigate the extent to which individualization of CT protocols is practiced across Norway. METHODS: This cross-sectional study involved collection of CT protocols and administration of a mini-questionnaire to obtain additional information about how CT examinations are individualized. All public hospitals performing CT to detect pulmonary embolism were invited, and 41% participated. RESULTS: Tailoring a standard protocol to different patient groups was more common than using dedicated protocols. Most of the available radiation dose-reduction approaches were used. However, implementation of these strategies was not systematic. Children and pregnant patients were examined without using dedicated CT protocols or by using protocol adjustments focusing on radiation dose reduction in 30% and 39% of the hospitals, respectively. CONCLUSION: Practice optimization is needed, especially the development of dedicated CT protocols or guidelines that tailor the existing protocol to pediatric and pregnant patients. Practice might benefit from a more systematic approach to individualization of CT examinations, such as inserting tailoring instructions into CT protocols.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7157970/

Record 876

pmcid: PMC7060706

pmid: 32071206

doi: 10.1073/pnas.1910003117

title: Integrated functional genomic analyses of Klinefelter and Turner syndromes reveal global network effects of altered X chromosome dosage.

authors

- Zhang X
- Hong D
- Ma S
- Ward T
- Ho M
- Pattni R
- Duren Z
- Stankov A
- Bade Shrestha S
- Hallmayer J
- Wong WH
- Reiss AL
- Urban AE

journal: Proceedings of the National Academy of Sciences of the United States of America

journalAbbr: Proc Natl Acad Sci U S A

year: 2020

pubdate: 2020 Mar 3

volume: 117

issue: 9

pages: 4864-4873

abstract: In both Turner syndrome (TS) and Klinefelter syndrome (KS) copy number aberrations of the X chromosome lead to various developmental symptoms. We report a comparative analysis of TS vs. KS regarding differences at the genomic network level measured in primary samples by analyzing gene expression, DNA methylation, and chromatin conformation. X-chromosome inactivation (XCI) silences transcription from one X chromosome in female mammals, on which most genes are inactive, and some genes escape from XCI. In TS, almost all differentially expressed escape genes are down-regulated but most differentially expressed inactive genes are up-regulated. In KS, differentially expressed escape genes are up-regulated while the majority of inactive genes appear unchanged. Interestingly, 94 differentially expressed genes (DEGs) overlapped between TS and female and KS and male comparisons; and these almost uniformly display expression changes into opposite directions. DEGs on the X chromosome and the autosomes are coexpressed in both syndromes, indicating that there are molecular ripple effects of the changes in X chromosome dosage. Six potential candidate genes (RPS4X , SEPT6 , NKRF , CX0rf57 , NAA10 , and FLNA) for KS are identified on Xq, as well as candidate central genes on Xp for TS. Only promoters of inactive genes are differentially methylated in both syndromes while escape gene promoters remain unchanged. The intrachromosomal contact map of the X chromosome in TS exhibits the structure of an active X chromosome. The discovery of shared DEGs indicates the existence of common molecular mechanisms for gene regulation in TS and KS that transmit the gene dosage changes to the transcriptome.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7060706/

Record 877

pmcid: PMC7571560

pmid: 33094019

doi: 10.1055/s-0040-1715174

title: Interdisciplinary Approach for the Medical Management of Gestational Gigantomastia.

authors

- Cabrera C
- Radolec M
- Prescott A
- De La Cruz C
- Beck S

journal: AJP reports

journalAbbr: AJP Rep

year: 2020

pubdate: 2020 Jul

volume: 10

issue: 3

pages: e304-e308

abstract: Background Gestational gigantomastia is a rare and debilitating condition that is thought to result from

hormone hypersensitivity. Several definitions have been proposed using breast weight and change in body mass index, but the breast growth is best summarized as rapid, diffuse, and excessive. Case We report a case of a 31-year-old woman with a history of infertility and cystic fibrosis that developed pathologic breast growth during hormonal preparation for in vitro fertilization. Her serum laboratories were unremarkable, and she was medically managed until 31 weeks of gestation. After delivery, she experienced rapid decrease in breast size and was followed by plastic surgery with plan to allow spontaneous regression with interval breast reduction Conclusion We highlight a successful interdisciplinary medical management approach, which helped to avoid a morbid, intrapartum breast reduction.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7571560/

Record 878

pmcid: PMC7590456

pmid: 33109161

doi: 10.1186/s12902-020-00639-9

title: Isoniazid-induced gynaecomastia: report of a paediatric case and review of literature.

authors

- Poon SWY
- Siu KK
- Tsang AMC

journal: BMC endocrine disorders

journalAbbr: BMC Endocr Disord

year: 2020

pubdate: 2020 Oct 27

volume: 20

issue: 1

pages: 160

abstract: BACKGROUND: Gynaecomastia is a fairly common condition in puberty but is rare in prepubertal boys. While it is necessary to exclude possible endocrinopathay in prepubertal gynaecomastia, medication is an important and potentially reversible cause to consider in new onset gynaecomastia. Isoniazid-induced gynaecomastia has been reported in adult males, but none was reported in the paediatric population and general paediatricians may not be aware of this uncommon side effect. CASE PRESENTATION: We hereby report a 11-year-old prepubertal boy who developed gynaecomastia while taking anti-tuberculosis drugs. Investigations excluded endocrinopathies. Gynaecomastia subsided 8 weeks after stopping isoniazid. CONCLUSION: This case is the first paediatric case report describing the association of gynaecomastia with isoniazid use. It is important for general paediatricians to recognize this entity, as prompt diagnosis and cessation of the offending drug can lead to resolution of the problem.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7590456/

Record 879

pmcid: PMC7546153

pmid: 33037517

doi: 10.1007/s40487-020-00132-2

title: Living with Advanced Hormone-Sensitive Prostate Cancer and Treatment with Abiraterone and Androgen Deprivation Therapy: The Patient, Nursing and Physician Perspective.

authors

- Collier T
- Smith S
- Greenwood M
- Ng K

journal: Oncology and therapy

journalAbbr: Oncol Ther

year: 2020

pubdate: 2020 Dec

volume: 8

issue: 2

pages: 197-207

abstract: This article is co-authored by a patient with metastatic hormone-sensitive prostate cancer who is receiving abiraterone and androgen deprivation therapy treatment in Manchester, UK. The patient relates his personal experiences struggling with the diagnosis, his experience with treatment and the physical, emotional and psychosexual impact on his life. After his diagnosis, the patient has become an outspoken advocate and fundraiser for prostate cancer awareness and wants to ensure that novel treatments with proven efficacy and tolerability, such as abiraterone, are available for all men in his condition. The specialist nursing and physician perspectives, provided by healthcare professionals based in London who are not directly involved in this patient's care, were written in response to the challenges and concerns highlighted by this patient. The role of the specialist nurse as a key healthcare professional in the cancer patient journey, particularly in managing the complex physical and emotional side effects of treatment, is highlighted in this perspective piece. The physician reviews the current difficulties of establishing an effective screening programme in prostate cancer, the common side effects of hormone treatment and the significant progress and challenges in novel drug development and prescription in metastatic hormone-sensitive prostate cancer. While written primarily from the perspective of a patient and healthcare professionals in England, many messages in this commentary would resonate with patients and professionals involved in the care of prostate cancer worldwide.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7546153/

Record 880

pmcid: PMC7728242

pmid: 33301485

doi: 10.1371/journal.pone.0243212

title: Long-term outcomes after gonadotropin-releasing hormone agonist treatment in boys with central precocious puberty.

authors

- Shim YS
- Lim KI
- Lee HS
- Hwang JS

journal: PloS one

journalAbbr: PLoS One

year: 2020

pubdate: 2020

volume: 15

issue: 12

pages: e0243212

abstract: OBJECTIVE: Gonadotropin-releasing hormone agonist (GnRHa) treatment improves the potential for gaining height in patients with central precocious puberty (CPP). However, most studies have focused on girls because CPP in boys is relatively rare. Therefore, we aimed to determine the effect of GnRHa treatment on auxological outcomes in boys with CPP. METHODS: Eighty-five boys with CPP were treated with leuprolide or triptorelin acetate 3.75 mg over 2 years. Anthropometry, bone age, sexual maturity rating, and predicted adult height (PAH) were assessed every 6 months. Furthermore, 20 boys were followed up after treatment discontinuation until achievement of the final adult height (FAH). RESULTS: The mean chronological age (CA) and bone age (BA) of the patients with CPP at treatment initiation were 9.5 ± 0.5 years and 11.7 ± 0.9 years, respectively. The mean duration of treatment was 2.87 ± 0.63 years. The PAH at treatment initiation was 172.1 cm (-0.23 ± 1.05 PAH standard deviation score). The PAH at treatment discontinuation (176.2 ± 6.6 cm) was significantly higher than the pretreatment PAH. In addition, the mean final adult height in the 20 boys who were followed up after discontinuation of treatment was 173.4 ± 5.8 cm, which was significantly higher than the initial PAH (170.1 ± 4.5 cm; p = 0.006). In multivariate analysis, the height gain (the difference between the FAH and PAH at treatment initiation) significantly correlated with the target height. CONCLUSION: Long-term GnRHa treatment significantly improved the growth potential and FAH in boys with CPP.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7728242/

Record 881

pmcid: PMC7294380

pmid: 32529879

doi: 10.1177/0300060520920463

title: Management of gestational gigantomastia with breast reconstruction after mastectomy: case report and literature review.

authors

- Qin F
- Si L
- Zhang H
- Zhang M
- Zeng A
- Long F
- Yu N
- Liu Z
- Wang X

journal: The Journal of international medical research

journalAbbr: J Int Med Res

year: 2020

pubdate: 2020 Jun

volume: 48

issue: 6

pages: 300060520920463

abstract: Gestational gigantomastia (GGM) is a rare complication of pregnancy. The etiology of GGM is yet to be fully established. Treatment methods for GGM include medical therapy and surgery. If medical treatment is unsuccessful, surgery may be required. Currently available surgical interventions are either breast reduction or mastectomy with delayed reconstruction. We report a case of a 25-year-old woman (G1P1) who presented with massive enlargement of both breasts during puerperium. Because of the limited effect of medical therapy,

surgical intervention was considered to be the first choice. Bilateral mastectomies with grafting of the nipple-areola complex and immediate bilateral tissue expander implantation were performed. Reconstruction was fully completed 8 months after the initial procedure by replacing tissue expanders with definitive implants. Despite being a benign condition, GGM can turn into a serious problem. GGM can be successfully reconstructed by mastectomy with delayed reconstruction and grafting of the nipple-areola complex.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7294380/

Record 882

pmcid: PMC7053411

pmid: 32190188

doi:

title: McCune Albright Syndrome.

authors

- Jibbe N
- Jibbe A
- Rajpara A

journal: Kansas journal of medicine

journalAbbr: Kans J Med

year: 2020

pubdate: 2020

volume: 13

issue:

pages: 49-50

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7053411/

Record 883

pmcid: PMC7187250

pmid: 31927774

doi: 10.1111/ijcp.13480

title: Men's Sexual Health Questionnaire score changes vs spontaneous sexual adverse event reporting in men treated with dutasteride/tamsulosin combination therapy for lower urinary tract symptoms secondary to benign prostatic hyperplasia: A post hoc analysis of a prospective, randomised, placebo-controlled study.

authors

- Roehrborn CG
- Rosen RC
- Manyak MJ
- Palacios-Moreno JM
- Wilson TH
- Lulic Z
- Giuliano F

journal: International journal of clinical practice

journalAbbr: Int J Clin Pract

year: 2020

pubdate: 2020 May

volume: 74

issue: 5

pages: e13480

abstract: AIM: To assess the impact of baseline characteristics on Men's Sexual Health Questionnaire (MSHQ) total scores and to evaluate the clinical relevance of MSHQ changes and their association with spontaneously reported sexual adverse events (SexAEs) in patients with benign prostatic hyperplasia. METHODS: This was a post hoc analysis of the Phase 4 FDC116115 study, in which patients aged ≥50 years were randomised 1:1 to receive a fixed-dose combination of dutasteride 0.5 mg and tamsulosin 0.4 mg (DUT-TAM FDC), or placebo. End-points included: change in MSHQ total scores by baseline characteristics and SexAEs; cumulative distribution function for change from baseline to month 12 in MSHQ total score and the ejaculation, erection, satisfaction and sexual desire (libido) domain scores; and relationship between changes in MSHQ scores and SexAEs. RESULTS: The intent-to-treat population comprised 489 patients (DUT-TAM FDC, n = 243; placebo, n = 246). The mean reduction in total MSHQ score was greater in patients with SexAEs across both groups, compared with patients without SexAEs. Most patients reporting any SexAE (86% DUT-TAM FDC, 67% placebo) had a worsening of the MSHQ total score at month 12 compared with baseline. Specifically, 90% (DUT-TAM FDC) and 75% (placebo) of patients reporting an ejaculation SexAE and 73% (DUT-TAM FDC) and 87% (placebo) of patients reporting an erection SexAE had a worsening of MSHQ ejaculation and erection domain scores, respectively, at month 12. A threshold effect for incident SexAE was observed; patients showing a decrease of approximately 6-10 points in the total MSHQ score were more likely to report SexAEs. CONCLUSION: Findings support the clinical utility of the MSHQ tool in assessing the impact of DUT-TAM on sexual function by linking numerical changes in MSHQ scores to spontaneously reported SexAEs for the first time. The threshold effect for incidence of SexAEs warrants further investigation to determine its clinical relevance.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7187250/

Record 884

pmcid: PMC7430475

pmid: 33015667

doi: 10.1080/26895269.2020.1749921

title: Orchiectomy in transgender individuals: A motivation analysis and report of surgical outcomes.

authors

- van der Sluis WB
- Steensma TD
- Bouman MB

journal: International journal of transgender health

journalAbbr: Int J Transgend Health

year: 2020

pubdate: 2020

volume: 21

issue: 2

pages: 176-181

abstract: Background: Different surgical procedures are available for transgender women wishing to undergo genital gender-affirming surgery. Aim: To assess preoperative motivations, the frequency of orchiectomy procedures, and postoperative outcomes of orchiectomy in transgender individuals. Methods: All transgender

individuals who underwent orchiectomy in the period between January 2012 and January 2020 at our institution were retrospectively identified. A chart study was conducted, recording motivations, demographics, perioperative characteristics and surgical outcomes. The frequency of orchiectomy and vaginoplasty procedures during the study period were determined and compared. Results: During the study period, an increase of performed orchiectomy procedures was observed. The orchiectomy/vaginoplasty ratio was 0.01-0.07 in the period 2012-2018 and 0.24 in 2019. A total of 43 transgender individuals were retrospectively identified. Sixteen (37%) initially wished to undergo a vaginoplasty, but did not meet institutional requirements (nonsmoker, BMI < 30kg/m 2) or were not eligible for vaginoplasty surgery because of interfering somatic or mental health issues. Fourteen (33%) individuals who underwent orchiectomy regarded it as a preceding step to a future vaginoplasty procedure. Out of these fourteen, one person is now on the waiting list for vaginoplasty surgery. Thirteen (30%) others did not report a desire to pursue vaginoplasty in the future. This was mostly motivated by the absence of genital dysphoria or motivations related to their gender identity, with a desire to discontinue anti-testosterone treatment. The postoperative course was uncomplicated in 39 (91%) individuals. Conclusion: In the Netherlands, especially in the last year, the frequency of orchiectomy procedures has increased. Reasons that people chose to undergo this procedure include: not being eligible for a vaginoplasty procedure, seeing it as a preceding step to a possible future vaginoplasty or other identity-related motivations (i.e., non-binary gender identification or absence of genital dysphoria).

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7430475/

Record 885

pmcid: PMC7062947

pmid: 31485954

doi: 10.1007/s12687-019-00435-6

title: Perspectives of adults with Klinefelter syndrome, unaffected adolescent males, and parents of affected children toward diagnosis disclosure: a Thai experience.

authors

- Suwannachat S
- Wattanasirichaigoon D
- Arunakul J
- Chirdkiatgumchai V
- Tim-Aroon T

journal: Journal of community genetics

journalAbbr: J Community Genet

year: 2020

pubdate: 2020 Apr

volume: 11

issue: 2

pages: 171-181

abstract: Little is known about disclosure of diagnosis to males with Klinefelter syndrome (KS) diagnosed before birth or during childhood, especially in Asian context. Insufficient preparation for disclosure communication could lead to the child's depression, anxiety, or disrupted self-esteem. This is an interventional and qualitative interview study with a brief self-reported survey. The study consists of three phases. In phase I, adults with KS were asked about their experiences and viewpoints toward KS disclosure and diagnosis through individual interviews. In phase II, unaffected adolescent males were given information about KS and then asked to give hypothetical answers to a questionnaire followed by a group interview, as if they had KS. In phase III, parents of children with KS were asked to complete a questionnaire followed by a group interview which included a discussion about their disclosure plans and their concerns. The parents were provided information obtained from adults with KS and unaffected adolescent males, and were then interviewed again as a group. We found

that the most hurtful issues were discovering their infertility from KS and late disclosure. From the perspective of unaffected adolescent males, early disclosure in early or mid-teens in a neutral, supportive, and relaxed manner, along with information on benefits of hormonal treatment, the assurance of biological sex as "male," and avoidance of some sensitive words are strongly recommended, as a way to promote self-confidence and positive coping with the diagnosis. After intervention, the parents reported more confidence and less anxiety about disclosing the diagnosis to their children, and indicated that they would disclose KS at an earlier age. For unaffected adolescent males, though it is a hypothetical scenario and their reaction may differ from affected adolescents, unaffected adolescent males' viewpoints on how they might react are more or less representative of cultural changes for the new generation, especially in the Asian context. In addition, their viewpoints influenced parents' decision and genetic counseling plans for the diagnosis disclosure of KS to their children. We propose a preliminary guideline for KS diagnosis disclosure.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7062947/

Record 886

pmcid: PMC7469722

pmid: 32865090

doi: 10.1177/0300060520939672

title: Physical deviation and precocious puberty among school-aged children in Leshan City: an investigative study.

authors

- Wei Q
- Wu M
- Li YL
- Rao R
- Li S
- Cen Q
- Wu H
- Lv L
- Huang M
- Ge YP
- Lu ZL
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- Cao Y
- Liu TT
- Wang L

journal: The Journal of international medical research

journalAbbr: J Int Med Res

year: 2020

pubdate: 2020 Aug

volume: 48

issue: 8

pages: 300060520939672

abstract: OBJECTIVE: We investigated physical deviation and precocious puberty among school-aged children in Leshan City, to provide a theoretical basis for the management of precocious puberty in children. METHODS: We selected 12 primary schools of Leshan City using a cluster random sampling method and conducted

physical examinations among healthy students aged 4 12 years. A total of 11,000 students were recruited (5502 boys and 5498 girls). We measured body mass index (BMI), and participants were tested for precocious puberty according to the Tanner stages and standard maps. Nutritional status was also evaluated. RESULTS: Obese and overweight children accounted for a high proportion of participants; the prevalence of underweight was the lowest. The prevalence of obesity among boys was higher than that in girls. Precocious puberty was mainly observed in girls, particularly those age 7 years old. The prevalence of precocious puberty among overweight and obese children was higher than that in children with normal weight. CONCLUSION: We identified a significant sex difference in precocious puberty among children in Leshan City. Overweight and obesity may be associated with precocious puberty.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7469722/

Record 887

pmcid: PMC7732732

pmid: 33324515

doi: 10.7759/cureus.11432

title: Post Reduction Mammaplasty Pyoderma Gangrenosum: An Unusual Presentation of a Misdiagnosed Entity.

authors

- Zapata Alvarez J
- Patrón Gómez A

journal: Cureus

journalAbbr: Cureus

year: 2020

pubdate: 2020 Nov 11

volume: 12

issue: 11

pages: e11432

abstract: Pyoderma gangrenosum is a rare skin disorder that could be triggered after surgical trauma. However, more uncommon, this condition could follow breast surgery. An opportune diagnosis and adequate treatment are essential in order to avoid wound chronicity and aesthetic sequels. We report a 51-year-old woman who presented multiple wound complications after a reduction mammaplasty. This particular case is unique, because bilateral and unusual presentation after another uncomplicated previous surgical procedure, and illustrates how a patient could be taken through different rational surgical therapeutic modalities without improvement, and deterioration of clinical picture. Finally, a diagnosis of Pyoderma gangrenosum was established, and immunosuppressive treatment was completed, with an appropriate resolution of this problematic condition.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7732732/

Record 888

pmcid: PMC7590426

pmid: 33133250

doi: 10.3892/ol.2020.12177

title: Predictive value of T2-weighted magnetic resonance imaging for the prognosis of patients with mass-type breast cancer with peritumoral edema.

authors

- Liang T

- Hu B
- Du H
- Zhang Y

journal: Oncology letters

journalAbbr: Oncol Lett

year: 2020

pubdate: 2020 Dec

volume: 20

issue: 6

pages: 314

abstract: The aim of the present study was to investigate the role of edema surrounding breast cancer masses in the prognostic prediction of magnetic resonance imaging (MRI) T2-weighted fat suppression sequence. For this purpose, 80 patients with mass-type breast cancer underwent conventional plain breast MRI, dynamic contrast-enhanced (DCE)-MRI or diffusion-weighted MRI scan. The associations between edema around the mass on MRI T2 fat suppression sequence plain scan and tumor stage, pathological findings, immunohistochemical findings and axillary lymph node metastasis were analyzed. The results revealed the presence of edema around the mass on the MRI T2 fat suppression sequence plain scan in 35 patients. By contrast, there was no abnormal enhancement on the DCE-MRI, and the apparent diffusion coefficient value did not decrease in these areas. Compared with the remaining 45 patients, the 35 patients with peritumoral edema exhibited a higher tumor stage and a higher rate of axillary lymph node metastasis (all $P<0.05$). However, there was no significant difference in pathological classification or the expression of estrogen receptor, progesterone receptor and human epidermal growth factor receptor 2 (as determined by immunohistochemistry) between the two groups. In total, 12 cases of tumor shrinkage during neoadjuvant chemotherapy were accompanied by an improvement in edema. Taken together, the findings of the present study indicated that the presence of edema around the mass on the MRI T2 fat suppression sequence may predict poor prognosis in patients with mass-type breast cancer. Furthermore, the improvement of the peritumoral edema post-neoadjuvant chemotherapy may also be a predictor of a more favorable prognosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7590426/

Record 889

pmcid: PMC7025451

pmid: 32074227

doi: 10.1590/1984-0462/2020/38/2018294

title: PREPUBERTAL GYNECOMASTIA: A RARE MANIFESTATION OF MYOTONIC DYSTROPHY TYPE 1.

authors

- Miranda PSF
- Pereira EPMN
- Barreto JSCB
- Henriques MMV
- Mirante MASC
- Ramos LMJFC

journal: Revista paulista de pediatria : orgao oficial da Sociedade de Pediatria de Sao Paulo

journalAbbr: Rev Paul Pediatr

year: 2020

pubdate: 2020

volume: 38

issue:

pages: e2018294

abstract: OBJECTIVE: To present a case of bilateral gynecomastia in a prepubertal boy with autism spectrum disorder, diagnosed with myotonic dystrophy type 1. CASE DESCRIPTION: A 12-year-old boy with autism spectrum disorder presented at a follow-up visit with bilateral breast growth. There was a family history of gynecomastia, cataracts at a young age, puberty delay, and myotonic dystrophy type 1. The physical examination showed that he had bilateral gynecomastia with external genitalia Tanner stage 1. Neurologic examination was regular, without demonstrable myotonia. The analytical study revealed increased estradiol levels and estradiol/testosterone ratio. After excluding endocrine diseases, the molecular study of the dystrophia myotonica protein kinase gene confirmed the diagnosis of myotonic dystrophy type 1. COMMENTS: A diagnosis of prepubertal gynecomastia should include an investigation for possible underlying diseases. This case report highlights the importance of considering the diagnosis of myotonic dystrophy type 1 in the presence of endocrine and neurodevelopmental manifestations.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7025451/

Record 890

pmcid: PMC7527615

pmid: 33076198

doi: 10.1016/j.ijscr.2020.09.088

title: Primary chest wall tuberculosis mimicking gynecomastia: A case report.

authors

- Kakamad FH
- Hassan MN
- Salih AM
- Ahmed GS
- Abdullah BA
- Mohammed SH

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2020

pubdate: 2020

volume: 75

issue:

pages: 473-475

abstract: INTRODUCTION: Musculoskeletal tuberculosis (TB) is a rare variant of the disease. Involvement of the chest wall is even rarer. This paper aims to report a case of primary chest wall TB mimicking gynecomastia. CASE REPORT: An 11-year-old male presented with gradual left breast enlargement for one year duration, clinically diagnosed as a case of gynecomastia. On examination; there was a firm swelling involving left anterior chest wall elevating the nipple and areolar region. Ultrasound showed thick wall cystic lesion with internal debris and bone erosion. Computed tomography scan (CT scan) of the chest revealed a cystic lesion containing fluid with a similar cystic lesion in the substernal area. Under general anesthesia, through an anterolateral incision, a thick wall cystic lesion with a very thick pus content was found connecting to another similar lesion in the anterior mediastinum with a localized thickening of the pleura. Both of the lesions, and the fifth rib with a part of the fourth rib were resected and sent for histopathological examination which revealed multiple granulomas with caseating material, typical for tuberculosis. DISCUSSION: Pathogenesis of chest wall TB has been explained by

one or more of the three mechanisms: due to direct extension from an underlying disease; hematogenous dissemination, and direct extension from neighboring affected lymph node groups in the chest wall. CONCLUSION: Chest wall TB is a rare condition. It can mimic other pathologies due to nonspecific signs and symptoms. Surgical debridement with postoperative chemotherapy is the most effective strategy of management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7527615/

Record 891

pmcid: PMC7683364

pmid: 32961512

doi: 10.1016/j.clinimag.2020.08.008

title: Primary lymphoma of the breast: A report of two cases.

authors

- Gluskin J
- D'Alessio D
- Kim AC
- Morris EA
- Chiu A
- Noy A

journal: Clinical imaging

journalAbbr: Clin Imaging

year: 2020

pubdate: 2020 Dec

volume: 68

issue:

pages: 295-299

abstract: Primary breast lymphoma (PBL) should be distinguished from secondary breast lymphoma arising in the setting of lymphoma elsewhere in the body. Multimodality imaging is key to diagnosing PBL, and imaging manifestations thereof may indicate PBL and alter the treatment course. Treatment options including chemotherapy, radiation therapy, and/or surgery depend on histology. We report two cases of PBL, illustrating the transformative impact that multimodality imaging may have on clinical management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7683364/

Record 892

pmcid: PMC7586876

pmid: 33139983

doi: 10.5582/irdr.2020.03046

title: Primary neuroendocrine carcinoma of the breast: a rare presentation and review of the literature.

authors

- Salemis NS

journal: Intractable & rare diseases research

journalAbbr: Intractable Rare Dis Res

year: 2020

pubdate: 2020 Nov

volume: 9

issue: 4

pages: 233-246

abstract: Primary neuroendocrine carcinoma of the breast (NEBC) is a very rare occurrence accounting for less than 0.1% of all breast cancers. Typically, the tumor presents with ER- and PgR-positive and HER-2-negative status. Despite its luminal type, NEBC is associated with a more aggressive clinical course and poorer prognosis compared to the other types of invasive breast cancer. Clinical and radiological findings are nonspecific. The most common clinical manifestation is a palpable mass whereas in mammography the tumor most commonly appears as a round or oval mass without spiculated margins. Herein, a very rare case of NEBC is described in an asymptomatic patient who presented with an area of architectural distortion and the presence of microcalcifications that was incidentally detected on a screening mammography. A review of the literature has also been conducted. The diagnosis of NEBC requires a thorough investigation to exclude the possibility of a metastatic neuroendocrine tumor from another site because the two entities require different treatment approaches. Due to the rarity of the disease, the optimal therapeutic approach has not been clearly defined. Surgical resection is the mainstay of treatment. Further research is needed to better understand the molecular characteristics of NEBC and identify novel targeted therapies.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7586876/

Record 893

pmcid: PMC7534527

pmid: 33088017

doi: 10.1297/cpe.29.179

title: Relapsing 6q24-related transient neonatal diabetes mellitus with insulin resistance: A case report.

authors

- Uchida N
- Ohnishi T
- Kojima T
- Takahashi T
- Makita Y
- Fukami M
- Shibata H
- Hasegawa T
- Ishii T

journal: Clinical pediatric endocrinology : case reports and clinical investigations : official journal of the Japanese Society for Pediatric Endocrinology

journalAbbr: Clin Pediatr Endocrinol

year: 2020

pubdate: 2020

volume: 29

issue: 4

pages: 179-182

abstract: The overexpression of imprinted genes on chromosome 6q24 causes 6q24-related transient neonatal diabetes mellitus (6q24-TNDM). Most cases of 6q24-TNDM show transient diabetes mellitus (DM) during the neonatal period, followed by relapse after puberty. These two courses of DM are both characterized by insulin

insufficiency. However, there has been no previously reported case of 6q24-TNDM with insulin resistance at relapse. We report the case of a 10-yr-old Japanese girl with relapsing 6q24-TNDM. In the neonatal period, she had hyperglycemia and was treated with insulin injection until 2 mo of age. After several years of remission of DM, her HbA1c level increased to 7.4% at 10 yr of age. Homeostasis model assessment of insulin resistance (HOMA-IR) score was high at 6.2. After starting metformin therapy, her glycemic control improved along with normalization of HOMA-IR score. Using microsatellite marker analysis on the 6q24 region and array comparative genome hybridization, we diagnosed her with 6q24-TNDM due to paternally inherited duplication of 6q24. These data indicate that patients with 6q24-TNDM can develop relapsing DM with insulin resistance.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7534527/

Record 894

pmcid: PMC7136506

pmid: 32252213

doi: 10.6065/apem.2020.25.1.24

title: Relationship between final adult height and birth weight after gonadotropin-releasing hormone agonist treatment in girls with central precocious puberty.

authors

- Cho AY
- Ko SY
- Lee JH
- Kim EY

journal: Annals of pediatric endocrinology & metabolism

journalAbbr: Ann Pediatr Endocrinol Metab

year: 2020

pubdate: 2020 Mar

volume: 25

issue: 1

pages: 24-30

abstract: PURPOSE: The clinical significance of birth weight relative to gestational age in girls with central precocious puberty is unclear. This study sought to compare clinical parameters such as final adult height (FAH) and menarche onset after treatment with gonadotropin-releasing hormone agonist (GnRHa) on birth weight in girls with central precocious puberty treated. METHODS: This retrospective study reviewed data of 69 girls with precocious puberty who had reached their FAH in a long-term trial of GnRHa treatment between January 2007 and December 2017. The subjects were divided into small for gestational age (SGA) (n=19) and appropriate for gestational age (AGA) (n=50) groups. RESULTS: When starting GnRHa treatment, bone age was 10.9±0.9 and 10.3±0.8 years in the SGA and AGA groups, respectively (P<0.05). The predicted adult height (PAH) (established according to the Bayley-Pinneau average table) and advanced PAH (established according to the Bayley-Pinneau advanced table) were 151.5±4.8 cm and 155.8±4.9 cm in the SGA group, respectively, and 153.4±5.3 cm and 159.0±6.0 cm in the AGA group. After treatment, no significant difference in bone age was found between the groups. The time to menarche after treatment was 12.5±7.6 and 21.1±12.3 months in the SGA and AGA groups, respectively (P<0.05). FAH in the SGA and AGA groups was 161.0±4.7 cm and 161.6±5.0 cm, respectively, without a significant difference. CONCLUSION: SGA girls with precocious puberty have increased bone age and earlier menarche relative to AGA girls. However, no difference in FAH after treatment was found between these groups.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7136506/

Record 895

pmcid: PMC7190170

pmid: 32302297

doi: 10.1371/journal.pgen.1008666

title: Relaxed constraint and functional divergence of the progesterone receptor (PGR) in the human stem-lineage.

authors

- Marinić M
- Lynch VJ

journal: PLoS genetics

journalAbbr: PLoS Genet

year: 2020

pubdate: 2020 Apr

volume: 16

issue: 4

pages: e1008666

abstract: The steroid hormone progesterone, acting through the progesterone receptor (PR), a ligand-activated DNA-binding transcription factor, plays an essential role in regulating nearly every aspect of female reproductive biology. While many reproductive traits regulated by PR are conserved in mammals, Catarrhine primates evolved several derived traits including spontaneous decidualization, menstruation, and a divergent (and unknown) parturition signal, suggesting that PR may also have evolved divergent functions in Catarrhines. There is conflicting evidence, however, whether the progesterone receptor gene (PGR) was positively selected in the human lineage. Here we show that PGR evolved rapidly in the human stem-lineage (as well as other Catarrhine primates), which likely reflects an episode of relaxed selection intensity rather than positive selection. Coincident with the episode of relaxed selection intensity, ancestral sequence resurrection and functional tests indicate that the major human PR isoforms (PR-A and PR-B) evolved divergent functions in the human stem-lineage. These results suggest that the regulation of progesterone signaling by PR-A and PR-B may also have diverged in the human lineage and that non-human animal models of progesterone signaling may not faithfully recapitulate human biology.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7190170/

Record 896

pmcid: PMC7541972

pmid: 33029964

doi: 10.3988/jcn.2020.16.4.586

title: Reliability and Validity of the Korean Version of the Spinal and Bulbar Muscular Atrophy Functional Rating Scale.

authors

- Oh SI
- Oh J
- Park D
- Son K
- Park JS

journal: Journal of clinical neurology (Seoul, Korea)

journalAbbr: J Clin Neurol

year: 2020

pubdate: 2020 Oct

volume: 16

issue: 4

pages: 586-591

abstract: BACKGROUND AND PURPOSE: The Spinal and Bulbar Muscular Atrophy Functional Rating Scale (SBMAFRS) is a reliable and valid instrument for evaluating the functional status of patients with spinal and bulbar muscular atrophy (SBMA). This study aimed to validate a Korean version of the SBMAFRS in an SBMA population. METHODS: We applied the SBMAFRS to 64 SBMA patients at their regular follow-up clinical visits. The patients underwent clinical evaluations that included the 6-minute walking test (6MWT), forced vital capacity (FVC), manual muscle test, and the Penetration-Aspiration Scale (PAS). To estimate the stability of the SBMAFRS, the scale was reapplied to a subset of 31 randomly selected patients within 4 weeks of the initial test. The convergent validity was evaluated, and correlations were examined between SBMAFRS items and the muscular force, the total and subscores on the Revised Amyotrophic Lateral Sclerosis Functional Rating Scale (ALSFRS-R), FVC, PAS score, age at onset, disease duration, and 6MWT results. RESULTS: The internal consistency of the scale was confirmed by a high Cronbach's alpha (total raw alpha=0.867, total standardized alpha=0.863). The test-retest reliability as assessed by Spearman's rho was also high. The total score and subscores of the SBMAFRS were strongly correlated with the respective items and subscores of the ALSFRS-R, respiratory function, and the 6MWT. CONCLUSIONS: We have performed a validation study of the Korean version of a disease-specific functional rating scale for SBMA patients. The SBMAFRS is a useful tool for clinical practice and as a potential outcome measure for Korean SBMA patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7541972/

Record 897

pmcid: PMC7522893

pmid: 33015285

doi: 10.1016/j.ijwd.2020.05.002

title: Retrospective analysis of adverse events with spironolactone in females reported to the United States Food and Drug Administration.

authors

- Wang Y
- Lipner SR

journal: International journal of women's dermatology

journalAbbr: Int J Womens Dermatol

year: 2020

pubdate: 2020 Sep

volume: 6

issue: 4

pages: 272-276

abstract: BACKGROUND: Spironolactone is an off-label acne treatment that is commonly prescribed due to its low cost, efficacy, and tolerability. OBJECTIVE: This study aimed to classify the most common adverse reactions associated with spironolactone in women of all ages and analyze the relative risk of hyperkalemia for different age groups. METHODS: The U.S. Food and Drug Administration Adverse Event Reporting System (FAERS) database was analyzed for common adverse reactions associated with female patients taking spironolactone. Reported hyperkalemia adverse events with spironolactone were further subdivided by age group. Google Trends was used to examine public interest, and Altmetric was used to quantitate scholarly mentions of spironolactone. Yearly data were compared with adverse events in the FAERS database. RESULTS: The most common adverse reaction in women taking spironolactone was hyperkalemia (16.1% of all

adverse events), but it was extremely uncommon in women age ≤45 years (1.9% of all hyperkalemia cases). Increased Google searches and scholarly mentions in the Altmetric database for spironolactone were also associated with increased reporting of adverse events in the FAERS database for men and women combined. CONCLUSION: Women taking spironolactone should be counseled that hyperkalemia is the most common adverse event but is uncommon in those age ≤45 years. Public and academic interest in spironolactone has increased in recent years, and although prescribing data are not available, this interest may account for the increased reporting to FAERS during the same time period.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7522893/

Record 898

pmcid: PMC7192691

pmid: 32367917

doi: 10.1055/s-0040-1709427

title: Retrospective Study of Prosthetic Augmentation Mammoplasty in Transwomen.

authors

- Balakrishnan TM
- Nagarajan S
- Jaganmohan J

journal: Indian journal of plastic surgery : official publication of the Association of Plastic Surgeons of India

journalAbbr: Indian J Plast Surg

year: 2020

pubdate: 2020 Mar

volume: 53

issue: 1

pages: 42-50

abstract: Introduction Hormonal therapy and gender-conforming surgeries are the treatments of choice in appropriately selected male-to-female transsexuals. The Prosthetic Augmentation Mammoplasty (PAM) in transwomen is the second most common sex conforming procedure performed in our department. Aim This study was aimed to assess the esthetic outcomes of PAM performed from 2007 to 2017 in 42 transwomen. Patients and Methods It is a retrospective study wherein the documents of 42 transwomen who had undergone PAM were reviewed. Twenty-three transwomen who had pinch fold thickness more than 2.5 cm had undergone subglandular/subfascial (SG/SF) plane augmentation; rest had dual plane augmentation. All the cases were followed-up for an average period of 45 months. A validated institutional score for subjective assessment and objective assessment (by two independent observers) were used at the end of follow-up period. Statistical Analysis Used Student's t -test was used for statistical analysis. Results On an average, 92.85% of transwomen achieved grade-A score with both subjective and objective assessment scoring system. Conclusion PAM gives a finishing and colossal touch for transwomen to orient their somatic sex to psychic sex. To get the best cosmetic outcomes and lasting results the PAM in transwomen has to be made a more reasoning and scientific procedure rather than an emotional and irrational procedure. Author's PAM approach facilitates transwomen to rejoice their veritable total genderness in harmony with their gender identity.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7192691/

Record 899

pmcid: PMC7319285

pmid: 32589678

doi: 10.1371/journal.pone.0235050

title: Silicone Induced Granuloma of Breast Implant Capsule (SIGBIC) diagnosis: Breast Magnetic Resonance (BMR) sensitivity to detect silicone bleeding.

authors

- Fleury EFC

journal: PloS one

journalAbbr: PLoS One

year: 2020

pubdate: 2020

volume: 15

issue: 6

pages: e0235050

abstract: OBJECTIVE: To evaluate the sensitivity (S) of BMRI to detect silicone gel bleeding in a prospective observational study, including consecutive patients referred for BMRI scan. METHODS: From January 2017 to March 2018, we evaluated patients with breast implants referred for BMRI in a prospective observational study. For SIGBIC diagnosis, we adopted three new original imaging features: black drop signal; T2* hyper signal mass; and delayed contrast enhancement, considered as irrevocable signs to detect gel bleeding (GB). Histology confirmed the presence of a silicone corpuscle in breast implant capsular specimens. The accuracy of BMRI SIGBIC findings to predict GB was determined. We also compared SIGBIC diagnosis criteria to those features proposed by the BI-RADS léxicon, considered as equivocal findings. RESULTS: 208 patients had SIGBIC diagnosis at BMRI, and the histology confirmed GB in all cases. There were no false-positive results. Compared to the BI-RADS equivocal findings (S = 0.74), SIGBIC criteria had better sensitivity for GB diagnosis. CONCLUSION: SIGBIC diagnosis has high sensitivity to predict GB by the three irrevocable BMRI features described by the authors. We suppose GB is underdiagnosed in clinical practice by BI-RADS features. TRIAL CERTIFICATION: Study protocol: Plataforma Brasil CAAE: 77215317.0.0000.0072.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7319285/

Record 900

pmcid: PMC7281099

pmid: 32438681

doi: 10.3390/cancers12051286

title: Spectrum of Germline BRCA1 and BRCA2 Variants Identified in 2351 Ovarian and Breast Cancer Patients Referring to a Reference Cancer Hospital of Rome.

authors

- Santonocito C
- Rizza R
- Paris I
- Marchis L
- Paolillo C
- Tiberi G
- Scambia G
- Capoluongo E

journal: Cancers

journalAbbr: Cancers (Basel)

year: 2020

pubdate: 2020 May 19

volume: 12

issue: 5

pages:

abstract: Pathogenic variants (PVs) carriers in BRCA1 or BRCA2 are associated with an elevated lifetime risk of developing breast cancer (BC) and/or ovarian cancer (OC). The prevalence of BRCA1 and BRCA2 germline alterations is extremely variable among different ethnic groups. Particularly, the rate of variants in Italian BC and/or OC families is rather controversial and ranges from 8% to 37%, according to different reports. By In Vitro Diagnostic (IVD) next generation sequencing (NGS)-based pipelines, we routinely screened thousands of patients with either sporadic or cancer family history. By NGS, we identified new PVs and some variants of uncertain significance (VUS) which were also evaluated in silico using dedicated tools. We report in detail data regarding BRCA1/2 variants identified in 517 out of 2351 BC and OC patients. The aim of this study was to report the incidence and spectrum of BRCA1/2 variants observed in BC and/or OC patients, tested in at Policlinico Gemelli Foundation Hospital, the origin of which is mainly from Central and Southern Italy. This study provides an overview of the variant frequency in these geographic areas of Italy and provides data that could be used in the clinical management of patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7281099/

Record 901

pmcid: PMC7473684

pmid: 32908833

doi: 10.12890/2020_001735

title: Spontaneous Breast Haematoma after Heparin Anticoagulation.

authors

- Gani I
- Kapoor R
- Saeed M

journal: European journal of case reports in internal medicine

journalAbbr: Eur J Case Rep Intern Med

year: 2020

pubdate: 2020

volume: 7

issue: 9

pages: 001735

abstract: UNLABELLED: Heparin is commonly used in clinical practice for the prevention and treatment of various thrombotic conditions. Its use can be associated with bleeding which can range from minor to life threatening. Non-traumatic causes of breast haematoma are very rare. We report a case of spontaneous bleeding into the breast in a female patient who was anticoagulated with heparin. LEARNING POINTS: Anticoagulant use can be associated with the adverse effect of bleeding at various sites.Physicians should be cautious of such bleeding which can occur at unsuspected sites.Our patient developed spontaneous breast haematoma after unfractionated heparin anticoagulation, and was successfully managed with cessation of anticoagulation, protamine, desmopressin and blood transfusion.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7473684/

Record 902

pmcid: PMC7396908

pmid: 32774571

doi: 10.1016/j.radcr.2020.07.017

title: Synchronous breast implant associated anaplastic large cell lymphoma (BIA-ALCL) and silicone induced granuloma of breast implant capsule (SIGBIC): What to learn.

authors

- Fleury EFC

journal: Radiology case reports

journalAbbr: Radiol Case Rep

year: 2020

pubdate: 2020 Oct

volume: 15

issue: 10

pages: 1736-1742

abstract: We recently described silicone induced granuloma of breast implant capsule (SIGBIC) as an implant capsule illness related to intact silicone breast implants. The precursor to SIGBIC development is gel bleeding/ shedding from the implant shell/interior content. Currently, although the literature widely discussed the pathogenesis of breast implant-associated anaplastic large cell lymphoma (BIA-ALCL), the trigger point for its development is still a black-box. In this case report, we report a 46-year-old woman with SIGBIC diagnosis in her right breast and BIA-ALCL in her left breast, diagnosed with ultrasound and breast magnetic resonance. Microscopy confirmed silicone bleeding from the implant surface/ content. The imaging findings reported that SIGBIC and BIA-ALCL were similar; however, BIA-ALCL had an intracapsular collection.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7396908/

Record 903

pmcid: PMC7203570

pmid: 32378360

doi: 10.1002/prp2.585

title: The hydroxypropyl-β-cyclodextrin-minoxidil inclusion complex improves the cardiovascular and proliferative adverse effects of minoxidil in male rats: Implications in the treatment of alopecia.

authors

- Maqoud F
- Zizzo N
- Mele A
- Denora N
- Passantino G
- Scala R
- Cutrignelli A
- Tinelli A
- Laquintana V
- la Forgia F
- Fontana S
- Franco M
- Lopedota AA

- Tricarico D

journal: Pharmacology research & perspectives

journalAbbr: Pharmacol Res Perspect

year: 2020

pubdate: 2020 Jun

volume: 8

issue: 3

pages: e00585

abstract: The efficacy of minoxidil (MXD) ethanolic solutions (1%-5% w/v) in the treatment of androgenetic alopecia is limited by adverse reactions. The toxicological effects of repeated topical applications of escalating dose (0.035%-3.5% w/v) and of single and twice daily doses (3.5% w/v) of a novel hydroxypropyl-β-cyclodextrin MXD GEL formulation (MXD/HP-β-CD) and a MXD solution were investigated in male rats. The cardiovascular effects were evaluated by telemetric monitoring of ECG and arterial pressure in free-moving rats. Ultrasonographic evaluation of cardiac morphology and function, and histopathological and biochemical analysis of the tissues, were performed. A pharmacovigilance investigation was undertaken using the EudraVigilance database for the evaluation of the potential cancer-related effects of topical MXD. Following the application of repeated escalating doses of MXD solution, cardiac hypertrophy, hypotension, enhanced serum natriuretic peptides and K + -ion levels, serum liver biomarkers, and histological lesions including renal cancer were observed. In addition, the administration of a twice daily dose of MXD solution, at SF rat vs human = 311, caused reductions in the systolic, diastolic, and mean blood pressure of the rats (-30.76 ± 3%, -28.84 ± 4%, and -30.66 ± 5%, respectively, vs the baseline; t test P < .05). These effects were not reversible following washout of the MXD solution. Retrospective investigation showed 32 cases of cancer associated with the use of topical MXD in humans. The rats treated with MXD HP-β-CD were less severely affected. MXD causes proliferative adverse effects. The MXD HP-β-CD inclusion complex reduces these adverse effects.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7203570/

Record 904

pmcid: PMC7023968

pmid: 32071573

doi: 10.1055/s-0039-3402073

title: The Science of Fat Grafting.

authors

- Shih L
- Davis MJ
- Winocour SJ

journal: Seminars in plastic surgery

journalAbbr: Semin Plast Surg

year: 2020

pubdate: 2020 Feb

volume: 34

issue: 1

pages: 5-10

abstract: Autologous fat grafting has become a popular and well-established technique used by plastic surgeons in a variety of aesthetic and reconstructive procedures. An understanding of the basic science principles underlying fat grafting is crucial to explaining its extensive utility for soft tissue rejuvenation, volume

augmentation, and body contouring-and the unpredictable fat resorption rates after grafting that pose a significant challenge for plastic surgeons. While the scientific principles of fat grafting can theoretically be exploited to optimize fat grafting techniques and increase fat tissue survival, a consensus has yet been established as to the best practices for this procedure. This review discusses the biology of adipose tissue and the scientific principles behind its behavior and survival in autologous fat grafting.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7023968/

Record 905

pmcid: PMC11132315

pmid: 32568802

doi: 10.1097/GRF.0000000000000549

title: Transgender Men and the Gynecologist.

authors

- Mayhew AC
- Cohen A
- Gomez-Lobo V

journal: Clinical obstetrics and gynecology

journalAbbr: Clin Obstet Gynecol

year: 2020

pubdate: 2020 Sep

volume: 63

issue: 3

pages: 588-598

abstract: Recently, greater attention has been paid to the care of gender dysphoric and gender incongruent individuals. Gynecologists may be called upon to care for individuals who were assigned female at birth throughout or following social, medical, or surgical gender transition. Thus, gynecologists need to be aware of language regarding sex and gender, treatment typically used for the care of gender dysphoric or incongruent individuals, and aspects of well gynecologic care necessary for these individuals. This review highlights these aspects of care for transgender males to aid the general gynecologist in the care and treatment of these individuals.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11132315/

Record 906

pmcid: PMC7438851

pmid: 32903708

doi: 10.3389/fpsyt.2020.00761

title: Treating Schizophrenia: Open Conversations and Stronger Relationships Through Psychoeducation and Shared Decision-Making.

authors

- Mucci A
- Kawohl W
- Maria C
- Wooller A

journal: Frontiers in psychiatry

journalAbbr: Front Psychiatry

year: 2020

pubdate: 2020

volume: 11

issue:

pages: 761

abstract: Integrated pharmacological and psychosocial treatments, such as psychoeducation (PE) and shared decision-making (SDM), have been shown to significantly improve outcomes for people living with schizophrenia (PLWS). Underpinning the success of these interventions is a strong therapeutic relationship between PLWS, their carers, and their healthcare team. While many recognize the value of this relationship, implementation of the interventions necessary to facilitate its construction remain low. In this article, we identify the barriers to developing productive therapeutic relationships and explain how PE and SDM, taking into account cultural difference, can improve adherence to treatment, strengthen therapeutic relationships, and ultimately equip patients to achieve better functional outcomes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7438851/

Record 907

pmcid: PMC7237335

pmid: 32045591

doi: 10.1016/j.urology.2020.01.032

title: Treatment of Estrogen Levels in the Management of Hypogonadism: An Anonymous Survey of ISSM Members.

authors

- Butaney M
- Thirumavalavan N
- Balasubramanian A
- McBride JA
- Gondokusumo J
- Pastuszak AW
- Lipshultz LI

journal: Urology

journalAbbr: Urology

year: 2020

pubdate: 2020 May

volume: 139

issue:

pages: 104-109

abstract: OBJECTIVE: To assess the variability in management of estrogen levels in men treated with testosterone therapy (TTh). With the significant increase in the last 2 decades in the treatment of hypogonadism and the use of TTh, detailed guidelines for the management of estrogen levels are necessary. 1 MATERIALS AND METHODS: An anonymous survey was electronically distributed to the membership of the International Society for Sexual Medicine including questions on demographics, symptoms, and their approach to management of estrogen in patients on TTh. Chi-square test was used to determine associations. RESULTS: The response rate was 22.5% (489/2168). A total 62.4% of respondents indicated that they check serum estrogen at initial evaluation, but only 54.7% monitor levels in patients on TTh (P = .02). Fellowship-trained and

North American respondents were more likely to monitor patients (P < .05). A total of 69.4%, 47.7%, and 14.4% respondents prescribe antiestrogenic medications in symptomatic patients with elevated estrogen levels, for asymptomatic-elevated estrogen levels, and prophylactically, respectively. Academic respondents were more likely to prescribe an antiestrogen medication to symptomatic patients and prophylactically (P < .05). Anastrozole was the most common medication prescribed for symptomatic hyperestrogenemia (62.3%), but starting doses varied significantly, from 1 mg weekly to 1 mg daily. CONCLUSION: Approximately 50% of practitioners treating men with TTh monitor estrogen levels. Symptoms play a role in prescribing patterns and significant variability in aromatase inhibitors regimens exists. Increased monitoring of estrogen levels in men on TTh will facilitate an understanding of the symptoms, effects of high and low estrogen levels, and aid in standardization of research and therapy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7237335/

Record 908

pmcid: PMC7221220

pmid: 32357117

doi: 10.1177/0300060519896914

title: Turner syndrome with rapidly progressive puberty: a case report and literature review.

authors

- Yuan X
- Zhu Z

journal: The Journal of international medical research

journalAbbr: J Int Med Res

year: 2020

pubdate: 2020 May

volume: 48

issue: 5

pages: 300060519896914

abstract: This report describes a clinically rare and atypical case of 46,X,idic(X)(q21.32)/45,X-type Turner syndrome with rapidly progressive puberty development. After 11 months of treatment with recombinant human growth hormone (rhGH), the child's height increased. After 18 months of treatment with rhGH, the child showed secondary sex characteristics. The child was followed up for 1 year after the appearance of the secondary sex characteristics, and regular menses were still present. This case indicates that modern molecular biology techniques should be used rationally to further investigate the existence of X-chromosome translocations and occult chimeras to prevent misdiagnosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7221220/

Record 909

pmcid: PMC7220508

pmid: 32332608

doi: 10.1097/MD.0000000000019645

title: Two head positions for orotracheal intubation with the trachway videolight intubating stylet with manual in-line stabilization: A randomized controlled trial.

authors

- Chan WH
- Cheng CP

- Chiu YL
- Hsu YC
- Hu MH
- Huang GS

journal: Medicine

journalAbbr: Medicine (Baltimore)

year: 2020

pubdate: 2020 Apr

volume: 99

issue: 17

pages: e19645

abstract: BACKGROUND: The Trachway Videolight Intubating Stylet is a video-assisted system with a rigid but malleable intubating stylet that facilitates endotracheal intubation. Minimizing cervical spine movement with manual in-line stabilization is essential for patients with cervical spine injuries such as multiple trauma. However, the intubation time of the Trachway Videolight Intubating Stylet and complications associated with intubation in patients with manual in-line stabilization in the neutral-head and head-lift positions remain unclear. METHODS: Patients (20-80 years old) who were scheduled to undergo surgery that required general anesthesia with tracheal intubation were randomly allocated to either a neutral-head (n = 62) or a head-lift position (n = 62) group. Manual in-line stabilization was performed to limit cervical spine mobility. We aimed to evaluate orotracheal intubation time and success rate in these 2 positions with the Trachway Videolight Intubating Stylet. RESULTS: Intubation was faster in the head-lift than in the neutral-head position (20 ± 10 and 25 ± 13 seconds, respectively, P = .000); intubation was equally successful in the 2 positions (96.8% vs 96.8%). Responses to intubation did not differ between positions (heart rate, P = .142; visual analog scale scores for throat soreness, P = .54). The only significant predictor of intubation time was the body mass index in the head-lift position group (P = .005). CONCLUSIONS: Intubation using the Trachway Videolight Intubating Stylet with manual in-line stabilization is faster in the head-lift position, and therefore preferable. However, if the head-lift position is not suitable, the neutral-head position is a sensible alternative, with comparable intubation success rate, heart rate change, and postoperative throat soreness.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7220508/

Record 910

pmcid: PMC8051630

pmid: 33342834

doi: 10.4103/ajps.AJPS_57_17

title: Ultrasonographic review of pediatric breast masses among Nigerian children in a tertiary hospital.

authors

- Olarinoye-Akorede SA
- Ibinaiye PO
- Suleiman LB

journal: African journal of paediatric surgery : AJPS

journalAbbr: Afr J Paediatr Surg

year: 2020

pubdate: 2020 Jul-Dec

volume: 17

issue: 3 & 4

pages: 54-58

abstract: BACKGROUND: Breast masses occur infrequently in children and adolescents. Most of these masses have proved that benign and conservative approach is the management of choice. Consequently, imaging has become crucial. The knowledge of the ultrasonographic features of childhood and adolescent breast masses is needed for age appropriate medical care. AIM: The aim of this study was to describe and document the ultrasonographic spectrum of breast masses in children and adolescents (0-19) years seen in the Ahmadu Bello University teaching hospital, Zaria. MATERIALS AND METHODS: A 2-year retrospective review of breast sonograms of 25 consecutively presenting children and adolescents (3 males and 22 females) who had palpable breast masses. Ultrasound scans were performed with a Mindray Machine DC-8 using the linear transducer at 7.5-12 MHz transducer frequency. Histopathological confirmation of the solid masses was also obtained. The statistical analysis of the data collected was done using the SPSS software version 20 (SPSS Inc., Chicago, IL, USA). RESULTS: The patients were between 40 days and 19 years old (mean 14.8 years and standard deviation 4.1). The ultrasonographic findings were those of infections, benign tumoral lesions and pubertal endocrine changes. Overall, fibroadenoma was the most common mass, seen in 14 (56%) of the patients. Other findings were simple cysts, non-puerperal and puerperal mastitis, juvenile papillomatosis and normal glandular pubertal breast tissue. The three male patients had gynaecomstia, pseudogynaecomastia and cystic lymphangioma of the chest wall presenting as unilateral breast masses, respectively. There was no malignancy recorded in any of the patients. CONCLUSION: Ultrasonographically, benign masses predominate which is concordant with surgical findings. Familiarity with these features would obviate the need for unnecessary invasive procedures which should be reserved for only deserving cases.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8051630/

Record 911

pmcid: PMC7751968

pmid: 33347511

doi: 10.1371/journal.pone.0244373

title: Unforeseen uses of oral contraceptive pills: Exploratory study in Jordanian community pharmacies.

authors

- Barakat M
- Al-Qudah R
- Akour A
- Al-Qudah N
- Dallal Bashi YH

journal: PloS one

journalAbbr: PLoS One

year: 2020

pubdate: 2020

volume: 15

issue: 12

pages: e0244373

abstract: BACKGROUND: The use of oral contraceptive pills (OCPs) as a birth control method is very common worldwide. OCPs have many other labeled non-contraceptive indications, and as a result there is an associated risk of improper use, as with any other medications. This study was designed to assess the unforeseen improper uses of OCPs observed by community pharmacists in Jordan. METHOD: A cross-sectional study design was conducted using a self-administered survey. A convenience sample (n = 380) of Jordanian community pharmacists, were recruited through social media resources. The survey included multiple-choice and open-ended questions. Descriptive statistics and correlation analyses were completed using SPSS. RESULTS: More than half of the recruited pharmacists (55.3%) were female, and the mean age of the

participants was 32.58 ± 9.94. The majority of the pharmacists (85%) had good knowledge about the non-contraceptive indications of OCPs. About 53% of them confirmed their exposure to cases of the improper use of OCPs. About 67.5% of the pharmacists who confirmed exposure to such cases, reported the topical use of OCPs for the enhancement of hair growth. Around 15% of those pharmacists stated that OCPs were used to give negative results for addictive drug screening tests. In the event that the pharmacists suspected improper use, more than 90% suggested they would refrain from dispensing the pills. CONCLUSION: This study has spotlighted many unforeseen uses of OCPs in Jordan and highlighted the need for restricted national regulations on the monitoring of OCP prescription/selling patterns in Jordan by policymakers. Moreover, there is a need for the establishment of national educational programs for the Jordanian community regarding the safe proper use of OCPs.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7751968/

Record 912

pmcid: PMC7340187

pmid: 32666014

doi: 10.1210/jendso/bvaa074

title: Unmasked Testicular Seminoma During Use of Hormonal Transgender Woman Therapy: A Hidden hCG-Secreting Tumor.

authors

- Elshimy G
- Tran K
- Harman SM
- Correa R

journal: Journal of the Endocrine Society

journalAbbr: J Endocr Soc

year: 2020

pubdate: 2020 Jul 1

volume: 4

issue: 7

pages: bvaa074

abstract: Management of gender-affirming hormone therapy (HT) in transgender women includes surveillance of testosterone (T) levels. Failure of T to suppress, despite adherence to therapy, warrants additional investigations for unexpected sources of T or factors stimulating T secretion. Possible causes include T or gonadotropin production by an occult neoplasm. Testicular cancer is the most common malignancy affecting biological men aged between 15 and 35 years. Patients may be asymptomatic until tumor burden is high and/or metastatic. Hormone-producing tumors have rarely been reported in treated transgender women. Routine screening tests are recommended in a gender-incongruent person as per the 2017 Endocrine Society guidelines with measurement of T levels every 3 months initially to reach a goal of less than 50 ng/dL. Expectations should be discussed in detail with the transgender person since anticipated physical changes may not be notable for 6 to 18 months. We herein describe a case of a transgender woman who underwent standard HT including gonadotropin suppression with a gonadotropin-releasing hormone agonist, whose total T level failed to suppress. Testing revealed an elevated serum level of the beta subunit of human chorionic gonadotropin (β-hCG), diagnostic of an hCG-secreting testicular seminoma, as the underlying cause of unexpected T production. This case illustrates how easily a testicular cancer can remain unnoticed because it can be asymptomatic and the necessity to be alert to, and act on, anomalous laboratory results during treatment of a transgender person.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7340187/

Record 913

pmcid: PMC7493856

pmid: 32933589

doi: 10.1186/s13256-020-02472-z

title: Van Wyk-Grumbach syndrome and oligosyndactyly in a 6-year-old girl: a case report.

authors

- Egodawaththe NS
- Seneviratne SN
- Gunasekara S
- Amarasekara SM
- Weerasekara K

journal: Journal of medical case reports

journalAbbr: J Med Case Rep

year: 2020

pubdate: 2020 Sep 16

volume: 14

issue: 1

pages: 166

abstract: BACKGROUND: Van Wyk-Grumbach syndrome refers to the development of isosexual precocious pseudopuberty and multicystic enlarged ovaries in the presence of hypothyroidism and delayed bone age. It is a rare presentation of untreated hypothyroidism. The prepubertal response in Van Wyk-Grumbach syndrome is always isosexual and mediated by very high thyroid-stimulating hormone levels acting through the follicle-stimulating hormone receptors inducing a follicle-stimulating hormonal effect. Early recognition and thyroid hormone replacement can completely regress precocious puberty and ovarian enlargement, while improving the final height achievement. Oligosyndactly is a congenital bony abnormality and can manifest either as an isolated malformation or as a component of a syndromic diagnosis. However, development of hypothyroidism in children with this peculiar bony deformity has rarely been described in the medical literature, with the exception of Cenani-Lenz Syndactyly syndrome. CASE PRESENTATION: We report the case of a 6-year-old Sri Lankan girl who presented with a 2-day history of vaginal bleeding and exertional dyspnea. She had marked short stature (well below -3 standard deviations) with an upper segment to lower segment ratio of 1.47. This girl had isolated breast development of Tanner stage 2. She was diagnosed to have acquired hypothyroidism secondary to autoimmune thyroiditis and also had macrocytic anemia, pericardial effusion, gonadotropin-releasing hormone-independent precocious puberty with radiological evidence of pubertal changes in the uterus, and multicystic ovaries. Interestingly, she also had post-axial oligosyndactyly in both feet and right-sided clubfoot. The diagnosis of Van Wyk-Grumbach syndrome was made based on the clinical and laboratory features. Her symptoms were successfully managed with L-thyroxine therapy. CONCLUSIONS: Acquired hypothyroidism is a relatively common endocrine disorder among children and early recognition is important to prevent serious complications like Van Wyk-Grumbach syndrome. Sexual precocity with delayed bone age and stunting should direct our minds toward this unique diagnosis. It is always necessary to identify the other associated anomalies in addition to the primary diagnosis since these features may direct to a syndromic diagnosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7493856/

Record 914

pmcid: PMC7093266

pmid: 32203990

doi: 10.5999/aps.2019.01361

title: Volumetric change of the latissimus dorsi muscle after postoperative radiotherapy in immediate breast reconstruction with an extended latissimus dorsi musculocutaneous flap.

authors

- Park TS
- Seo JY
- Razzokov AS
- Choi JS
- Kim MW
- Lee JW
- Kim HY
- Jung YJ
- Choo KS
- Song KH
- Nam SB

journal: Archives of plastic surgery

journalAbbr: Arch Plast Surg

year: 2020

pubdate: 2020 Mar

volume: 47

issue: 2

pages: 135-139

abstract: BACKGROUND: This study aimed to determine the magnitude of volume reduction of the latissimus dorsi (LD) muscle after treatment using only postoperative radiotherapy (PORTx) in patients who underwent immediate breast reconstruction using an extended LD musculocutaneous (eLDMC) flap after partial mastectomy. METHODS: We retrospectively reviewed 28 patients who underwent partial mastectomy and an eLDMC flap, received only PORTx, and underwent chest computed tomography (CT) 7 to 10 days after surgery and 18±4 months after the end of radiotherapy, from March 2011 to June 2016. The motor nerve to the LD was resected in all patients. One plastic surgeon performed the procedures, and the follow-up period was at least 36 months (mean, 46.6 months). The author obtained LD measurements from axial CT views, and the measurements were verified by an experienced radiologist. The threshold for statistical significance was set at $P<0.05$. RESULTS: A statistically significant decrease in the LD volume was found after the end of PORTx (range, 61.19%-80.82%; mean, 69.04%) in comparison to the measurements obtained 7 to 10 days postoperatively ($P<0.05$). All cases were observed clinically for over 3 years. CONCLUSIONS: The size of an eLDMC flap should be determined considering an average LD reduction of 69% after PORTx. Particular care should be taken in determining the size of an eLDMC flap if the LD is thick or if it occupies a large portion of the flap.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7093266/

Record 915

pmcid: PMC7431205

pmid: 32710775

doi: 10.1210/clinem/dgaa485

title: What Accounts for the Increased Incidence of Gynecomastia Diagnosis in Denmark from 1998-2017?

authors

- Braunstein GD

journal: The Journal of clinical endocrinology and metabolism

journalAbbr: J Clin Endocrinol Metab

year: 2020

pubdate: 2020 Oct 1

volume: 105

issue: 10

pages: e3810-1

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7431205/

Record 916

pmcid: PMC6568146

pmid: 31258263

doi: 10.4103/jiaps.JIAPS_70_18

title: A Clinical Approach to Diagnosis of Ambiguous Genitalia.

authors

- Khanna K
- Sharma S
- Gupta DK

journal: Journal of Indian Association of Pediatric Surgeons

journalAbbr: J Indian Assoc Pediatr Surg

year: 2019

pubdate: 2019 Jul-Sep

volume: 24

issue: 3

pages: 162-169

abstract: Disorders of sex development (DSD) are a sensitive and stressful condition for the family as well as the treating physician to deal with. The main issue in managing such cases is sex assignment. The decision is influenced by the cultural background, the sex of rearing, clinical features, the biochemical parameters including hormonal studies, the imaging reports, parental preference, fertility potential, and the assessment of mental make-up of the child when possible. In third world countries, there is diagnostic dilemma as most children with DSD present late and a detailed-lengthy work-up often delay their definitive treatment. In this article, the authors try to identify the important clinical features in children presenting with various types of DSD, which may aid in making a quick provisional clinical diagnosis and expediting the diagnostic work-up. The data have been gathered from 38 years of experience of the senior author while managing about 1200 cases of DSD in the pediatric intersex clinic at the tertiary care level institute.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6568146/

Record 917

pmcid: PMC6746688

pmid: 31372741

doi: 10.1007/s00266-019-01467-6

title: A New Tool for Breast Anthropometric Measurements: Presentation and Validation for Women and Men.

authors

- Mikołajczyk M

- Kasielska-Trojan A
- Antoszewski B

journal: Aesthetic plastic surgery

journalAbbr: Aesthetic Plast Surg

year: 2019

pubdate: 2019 Oct

volume: 43

issue: 5

pages: 1160-1170

abstract: INTRODUCTION: Anthropometric measurements of breasts are crucial for planning surgical procedures; however, there are no practical solutions for their quick, digital performance. The aim of the study was to present and validate a self-designed web application BreastIdea (BI) designed for indirect breast anthropometry. METHODS: Ten male and 10 female volunteers had their chests measured directly according to the routine clinical practice. Then their chests were photographed in non-standardised conditions. Corresponding measurements were performed using BI. Accuracy and both relative and absolute reliability of BI measurements were investigated. RESULTS: Breast assessments using BI yielded highly accurate results and presented near-perfect precision when compared to direct anthropometric measurements of the breast. Indirect anthropometry eliminates the necessity to trace the body's curves, which usually introduces a bias to linear measurements. CONCLUSION: BI web application is a reliable tool for indirect breast measurements in a clinical setting, providing accurate results regardless of chest pathology and photograph standardisation. LEVEL OF EVIDENCE IV: This journal requires that authors assign a level of evidence to each article. For a full description of these Evidence-Based Medicine ratings, please refer to the Table of Contents or the online Instructions to Authors www.springer.com/00266 .

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6746688/

Record 918

pmcid: PMC6467042

pmid: 31000949

doi: 10.4103/ijri.IJRI_317_18

title: A rare case of neonatal mastauxe: Case report.

authors

- Suthar B
- Aggarwal K

journal: The Indian journal of radiology & imaging

journalAbbr: Indian J Radiol Imaging

year: 2019

pubdate: 2019 Jan-Mar

volume: 29

issue: 1

pages: 89-93

abstract: Neonatal breast enlargements have varying number of presentations on imaging and these have been underreported in the literature. Our case report profiles a 2-week-old female who presented with a history of bilateral breast enlargement with redness and clear, non-bloody, milk-like discharge, who was clinically diagnosed and managed for neonatal mastitis, which was actually a neonatal breast enlargement only. Awareness on neonatal breast enlargement that can be just physiological without any associated mastitis or

inflammatory/infective changes can prevent unnecessary hospitalization and treatment with parenteral antibiotics.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6467042/

Record 919

pmcid: PMC6305890

pmid: 30671553

doi: 10.1016/j.ejvssr.2018.10.002

title: A Rare Cause of Upper Limb Lymphoedema.

authors

- Pinho ANMBM
- Ferreira JMM
- Carneiro AFL

journal: EJVES short reports

journalAbbr: EJVES Short Rep

year: 2019

pubdate: 2019

volume: 42

issue:

pages: 8-11

abstract: This is the first case report of upper extremity lymphoedema caused by a thyroid tumour. An 87 year old female patient with grade 3 lymphoedema of the upper right extremity was admitted. The lymphoedema had developed over the course of two years. Investigation identified a thyroid tumour, which compressed the mediastinal structures at the right superior thoracic outlet, causing venous congestion, oedema, and lymphoedema. The patient underwent thyroidectomy which markedly improved the lymphatic oedema. Resolution of the compression mechanism was an effective treatment, despite the severity and chronicity of the initial presentation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6305890/

Record 920

pmcid: PMC6321810

pmid: 30202914

doi: 10.1093/aje/kwy193

title: Acetaminophen (Paracetamol) Exposure During Pregnancy and Pubertal Development in Boys and Girls From a Nationwide Puberty Cohort.

authors

- Ernst A
- Brix N
- Lauridsen LLB
- Olsen J
- Parner ET
- Liew Z
- Olsen LH

- Ramlau-Hansen CH

  journal: American journal of epidemiology

  journalAbbr: Am J Epidemiol

  year: 2019

  pubdate: 2019 Jan 1

  volume: 188

  issue: 1

  pages: 34-46

  abstract: This study explored the association between exposure to acetaminophen during pregnancy and pubertal development using data from 15,822 boys and girls in the longitudinal Puberty Cohort, nested within the Danish National Birth Cohort. Use of acetaminophen was reported 3 times during pregnancy and 6 months postpartum. In total, 54% of mothers indicated use at least once during pregnancy. Between 2012 and 2017, sons and daughters provided information on a wide range of pubertal milestones-including Tanner stages, axillary hair growth, and age at menarche or voice break and first ejaculation-every 6 months from 11 years of age until full sexual maturation. Data were analyzed using a regression model for interval-censored data, providing adjusted mean monthly differences in age at attaining the pubertal milestones according to intrauterine cumulative (weeks) and trimester-specific acetaminophen exposure. Our results suggested a tendency towards slightly earlier attainment of almost all studied markers of female pubertal development with increasing number of weeks of exposure (i.e., about 1.5-3 months earlier age at pubic hair, axillary hair, and acne development comparing unexposed with those prenatally exposed for more than 12 weeks). Male pubertal development had no strong association with acetaminophen exposure.

  url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6321810/

Record 921

pmcid: PMC6736488

pmid: 31464890

doi: 10.1097/MD.0000000000016456

title: Acupoint-tuina therapy promotes lactation in postpartum women with insufficient milk production who underwent caesarean sections.

authors

- Lu P
- Ye ZQ
- Qiu J
- Wang XY
- Zheng JJ

  journal: Medicine

  journalAbbr: Medicine (Baltimore)

  year: 2019

  pubdate: 2019 Aug

  volume: 98

  issue: 35

  pages: e16456

  abstract: Breast milk is recognized and strongly recommended by the World Health Organization (WHO) as the optimal feeding for all babies. Breastfeeding is associated with better nutritional and non-nutritional outcomes

when compared to formula feeding, and has proven health benefits to both infants and their mothers. This clinical research is to examine the feasibility and efficacy of Acupoint-Tuina therapy in treating postpartum women who underwent C-sections and suffered from insufficient milk production.The patients in the control group received standard medical care, while the patients in the Tuina group received Tuina therapy during the next 48 hours in addition to standard care, given once daily for 2 days. To evaluate the efficacy of Tuina therapy, patients of both groups were assessed for surface temperature of breasts, volume of breasts, volume of breast milk production, serum PRL level, and uterus recovery at various time points.Tuina therapy significantly increased the milk production when compared to the control group, for as much as 13-fold and 10-fold of that in the control group on the third and fourth postpartum days. In addition, Tuina therapy also significantly increased the full breast enlargement and the serum PRL level change, and decreased the breast surface temperature rise. Last but not the least, Tuina therapy also accelerated the post-surgery recovery of uterus.During the early postpartum days, Tuina therapy increases the milk production and promotes other physiological changes supporting lactation for postpartum women with C-section delivery and insufficient breast milk production. The novel intervention is warranted for further investigation and validation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6736488/

Record 922

pmcid: PMC6873727

pmid: 31768183

doi: 10.1186/s13633-019-0068-7

title: Adrenocortical adenoma in a Sudanese girl with Beckwith-Wiedemann syndrome.

authors

- Elnaw EAA
- Abdalla AR
- Abdullah MA

journal: International journal of pediatric endocrinology

journalAbbr: Int J Pediatr Endocrinol

year: 2019

pubdate: 2019

volume: 2019

issue:

pages: 6

abstract: BACKGROUND: We report a case of right adrenocortical adenoma in a girl with features suggestive of Beckwith Wiedemann syndrome to show the importance of tumor surveillance in patients with Beckwith Wiedemann syndrome. CASE PRESENTATION: A 4-years-old female with features suggestive of Beckwith-Wiedemann syndrome presented with 9 months history of virilization. Hormonal investigations results showed high levels of testosterone (2.3 ng/ml, normal values 0.1-0.4 ng/ml), and DHEAS (73 ng/ml normal values 1-6 ng/ml) with normal cortisol level. Computed tomography revealed a right adrenal mass. She underwent right adrenalectomy. Histopathological examination of the resected adrenal gland showed adrenocortical adenoma. Her postoperative evaluation showed a normal testosterone level. CONCLUSION: Adrenocortical neoplasms though rare in children are well documented in Beckwith-Wiedemann syndrome patients. So tumor surveillance protocol should be employed, even in a resource-limited setting for early tumor detection and a better outcome.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6873727/

Record 923

pmcid: PMC7150273

pmid:

doi: 10.1016/B978-0-12-801238-3.02387-4

title: Antiviral Agents.

authors

- Paintsil E
- Cheng YC

journal: Encyclopedia of Microbiology

journalAbbr: Encyclopedia of Microbiology

year: 2019

pubdate: 2019

volume:

issue:

pages: 176-225

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7150273/

Record 924

pmcid: PMC6606474

pmid: 31328108

doi: 10.21037/gs.2018.10.03

title: Application of the Lalonde (horizontal-only scar) breast reduction technique for correction of gynaecomastia in dark skinned patients.

authors

- Kazzazi F
- Malata CM

journal: Gland surgery

journalAbbr: Gland Surg

year: 2019

pubdate: 2019 Jun

volume: 8

issue: 3

pages: 287-293

abstract: Surgical correction of gynaecomastia correction sometimes necessitates skin reduction in addition to resection of glandular tissue and liposuction of the fat deposits. Many skin reduction techniques have been described but all suffer from very noticeable and often poor scars that can manifest as hypertrophic or keloid scars in patients with dark skin. Three large gynaecomastia patients undergoing a modification of the Lalonde "no vertical scar" breast reduction technique designed to reduce the extent of scarring were reviewed. The Lalonde "no vertical scar" technique was successfully applied to three patients of African extraction with acceptable postoperative scarring. A description of the technique and the cosmetic results are presented. The Lalonde technique of female breast reduction can be applied to the treatment of male breast enlargement in patients at high risk of scar complications. It provides an alternative to the more widely used concentric skin reduction techniques. To our knowledge, this is the first report in literature of the application of the Lalonde "no vertical scar" surgical technique to the treatment of gynaecomastia in patients with darkly pigmented skin.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6606474/

Record 925

pmcid: PMC6431559

pmid: 30612811

doi: 10.1016/j.jadohealth.2018.10.296

title: Bicalutamide as an Androgen Blocker With Secondary Effect of Promoting Feminization in Male-to-Female Transgender Adolescents.

authors

- Neyman A
- Fuqua JS
- Eugster EA

journal: The Journal of adolescent health : official publication of the Society for Adolescent Medicine

journalAbbr: J Adolesc Health

year: 2019

pubdate: 2019 Apr

volume: 64

issue: 4

pages: 544-546

abstract: PURPOSE: The purpose of the study was to describe the novel use of bicalutamide in transgender youth. METHODS: This is a retrospective review of patients with gender dysphoria followed in the pediatric endocrine clinic at Riley Hospital for Children. RESULTS: Of 104 patients with gender dysphoria, 23 male-to-female adolescents received bicalutamide 50 mg daily as a second-line puberty blocker after insurance company denial of a gonadotropin-releasing hormone analog. Six patients received estrogen concurrently. Of 13 patients treated exclusively with bicalutamide seen in follow-up, 84.6% had breast development within 6 months, the majority being ≥ Tanner stage III. CONCLUSIONS: Bicalutamide may be an alternative to gonadotropin-releasing hormone analogs in transgender male-to-female youth who are also ready to undergo physical transition.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6431559/

Record 926

pmcid: PMC7061678

pmid: 32158862

doi: 10.1016/j.jpra.2018.08.003

title: Bilateral pseudoangiomatous stromal hyperplasia in childhood gigantomastia: A challenge in reconstruction and management.

authors

- Morone I
- de Andrade GP
- Cardoso PS
- de Oliveira AC
- Clímaco F
- Medeiros J
- Franco D

journal: JPRAS open

journalAbbr: JPRAS Open

year: 2019

pubdate: 2019 Mar

volume: 19

issue:

pages: 106-110

abstract: The presence of mammary nodules during childhood and adolescence is somewhat unusual, generally consisting of benign lesions. Despite the range of possible diagnoses, they are generally similar in clinical terms. In the rare cases where these lesions constitute a case of gigantomastia, the differential diagnosis must be between juvenile fibroadenomatosis, phyllodes tumours and other even less common diseases of the mammary gland, such as Pseudoangiomatous Stromal Hyperplasia (PASH). This is caused by the exacerbated proliferation of mature fibroblasts and myofibroblasts, with the trigger factor still unknown. This study reports on a rare case of bilateral PASH in a young patient, describing its diagnosis and the surgical technique used.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7061678/

Record 927

pmcid: PMC6335371

pmid: 30746423

doi: 10.1002/mdc3.12680

title: Bobble-Head Doll Syndrome in a Child with Suprasellar Arachnoid Cyst.

authors

- Saracoglu M
- Kazan MS
- Ozak A
- Haspolat S

journal: Movement disorders clinical practice

journalAbbr: Mov Disord Clin Pract

year: 2019

pubdate: 2019 Jan

volume: 6

issue: 1

pages: 83-84

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6335371/

Record 928

pmcid: PMC6852609

pmid: 31745435

doi: 10.4102/sajhivmed.v20i1.1017

title: Breast abnormalities in adolescents receiving antiretroviral therapy.

authors

- Dunlop JL

- Slemming W
- Schnippel K
- Makura C
- Levin LJ
- Rayne S
- Vujovic M
- Firnhaber C

journal: Southern African journal of HIV medicine

journalAbbr: South Afr J HIV Med

year: 2019

pubdate: 2019

volume: 20

issue: 1

pages: 1017

abstract: BACKGROUND: Antiretrovirals, particularly efavirenz (EFV), have been shown to cause breast abnormalities in adults. Little is known about the prevalence of these adverse effects among adolescents receiving antiretroviral therapy (ART). OBJECTIVES: The aim of this article was to examine the extent of breast abnormalities in adolescents receiving ART and determine any clinical associations. METHODS: A retrospective record review describing breast conditions in adolescents receiving ART at three facilities in Johannesburg was conducted. Patients aged 10-19 years, who presented from January to December 2014, were included in the study. Analyses were conducted to determine whether EFV was associated with increased breast conditions. RESULTS: Of the 631 patient records reviewed, 37 (6%) had an abnormal breast event documented; with 24/37 (65%) being male patients. Patients with abnormal breast conditions were 1.5 years older than patients with normal breast development ($p < 0.0005$). Forty-one abnormal breast events were observed in 37 patients, with 20 described as gynaecomastia or lipomastia (49%). Of the 37 patients, 44% ($n = 19$) had concurrent generalised lipodystrophy. Of those with an abnormal breast event, 71% of patients had CD4 counts > 500 cells/μL and were virologically suppressed ($n = 29$). Those on EFV had a significantly higher prevalence of breast abnormalities compared to other regimens ($p = 0.016$). CONCLUSION: Of the studied patients, 6% had an abnormal breast condition. The use of EFV and increased age were associated with breast abnormalities in this population. Further research is needed to better understand the implications of this potential side effect.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6852609/

Record 929

pmcid: PMC6896197

pmid: 31661803

doi: 10.3390/cancers11111669

title: Breast Cancer during Pregnancy-Current Paradigms, Paths to Explore.

authors

- Alfasi A
- Ben-Aharon I

journal: Cancers

journalAbbr: Cancers (Basel)

year: 2019

pubdate: 2019 Oct 28

volume: 11

issue: 11

pages:

abstract: Breast cancer is the most common form of malignancy in pregnant women. The prevalence of pregnancy-associated breast cancer (PABC) is up to 0.04% of pregnancies and is expected to rise in developed countries. PABC represents a unique clinical scenario which requires a delicate balance of risks and benefits for both maternal and fetal well-being. Currently, there is paucity of data regarding the short- and long-term outcomes of in-utero exposure to anti-neoplastic agents. In general, when possible, treatment for PABC should follow the same guidelines as in non-pregnant patients. Surgery, including sentinel lymph node biopsy, is possible during all trimesters of pregnancy. Radiotherapy is contraindicated during pregnancy, although it might be considered in highly selected patients based on risk-benefit assessment. Evidence supports that administration of chemotherapy may be safe during the second and third trimesters, with cessation of treatment three weeks prior to expected delivery. Currently, hormonal therapy and anti-HER2 agents are contraindicated during pregnancy and should be postponed until after delivery. Prematurity is associated with worse neonatal and long-term outcomes, and thus should be avoided. While current data on the long-term effects of anti-neoplastic treatments are reassuring, grade of evidence is lacking, hence additional large prospective studies with long-term follow-up are essential to rule out any treatment-induced adverse effects.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6896197/

Record 930

pmcid: PMC6335972

pmid: 30713605

doi: 10.4081/oncol.2019.376

title: Breast cancer, human immunodeficiency virus and highly active antiretroviral treatment; implications for a high-rate seropositive region.

authors

- Chirkut S

journal: Oncology reviews

journalAbbr: Oncol Rev

year: 2019

pubdate: 2019 Jan 14

volume: 13

issue: 1

pages: 376

abstract: Sub-Saharan Africa is the region in the world with the most people infected with the human immunodeficiency virus (HIV). The incidence of breast cancer is also rising in the region. This transcript focusses on the burden of these two diseases when they converge in the same populace. This comprehensive literature review of the topic suggests a trend towards an increasing incidence of breast cancer in the HIV-infected population, and the rationale for such a tendency is hypothesized, especially in the context of the availability of highly active antiretroviral therapy. Besides the age at diagnosis, all other clinical characteristics appear to be similar in HIV-positive and HIV-negative breast cancer populations. Outcomes of the different treatment modalities for breast cancer in HIV-positive patients are also appraised and finally innovative areas of future research are suggested along with plausible recommendations.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6335972/

Record 931

pmcid: PMC6936743

pmid: 31774116

doi: 10.1093/humrep/dez230

title: Breast development and satisfaction in women with disorders/differences of sex development.

authors

- van de Grift TC
- Kreukels BPC

journal: Human reproduction (Oxford, England)

journalAbbr: Hum Reprod

year: 2019

pubdate: 2019 Dec 1

volume: 34

issue: 12

pages: 2410-2417

abstract: STUDY QUESTION: What are the levels of breast development and satisfaction in women with a Disorder/Difference of Sex Development (DSD)? SUMMARY ANSWER: Compared with normative data, women with DSD reached lower Tanner stages and reported less breast satisfaction. WHAT IS KNOWN ALREADY: Women with DSD may have chromosomal and hormonal variations that can impact typical breast development. While much emphasis is placed on genital development in this group, little is known about breast development, satisfaction and their association. STUDY DESIGN, SIZE, DURATION: Data collection was part of the cross-sectional European multicenter dsd-LIFE study. Fourteen recruiting sites included 1040 participants between February 2014 and September 2015. PARTICIPANTS/MATERIALS, SETTING, METHODS: A total of 695 female-identifying participants were included (Turner n = 332, 46,XY DSD n = 141 and congenital adrenal hyperplasia n = 222), with a median age of 28 years. Clinical (i.e. history of hormone and surgical treatments, Tanner breast examination) and patient-reported (i.e. breast satisfaction, relationship status, sexual satisfaction and experienced femininity) data was collected by independent trained research staff. The relationship between breast development, satisfaction and femininity was assessed. Control data on breast development and satisfaction in women without DSD was retrieved from the literature. MAIN RESULTS AND THE ROLE OF CHANCE: Of the 695 participants, 61% had received estrogen replacement and 51% puberty induction therapy, whereas 2% had received breast augmentation surgery. Approximately 65% of participants had reached Tanner breast stage 5, which is substantially less than the general population (90%). Breast satisfaction was lower than normative data as well ($P < 0.001$, Cohen's $d = 0.45$). Breast size and breast satisfaction were associated with feelings of femininity. LIMITATIONS, REASONS FOR CAUTION: Limitations include the sample representativeness (e.g. regarding the clinical heterogeneity) and the limited in-depth knowledge on (prior) hormonal regimens. Furthermore, no (matched) control data was collected as part of this study. WIDER IMPLICATIONS OF THE FINDINGS: In order to support the psychosexual well-being of women with DSD, enhancing breast development by sufficient hormone replacement and possible augmentation surgery is advocated. The scope of DSD management should be beyond genital development only and consider breasts as well. STUDY FUNDING/COMPETING INTEREST(S): The study was funded by the European Union Seventh Framework Program (FP7/2007-2013) under grant agreement no. 305 373. There are no competing interests. TRIAL REGISTRATION NUMBER: German Clinical Trials Register: Registration identification number: DRKS00006072.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6936743/

Record 932

pmcid: PMC6348285

pmid: 30701011

doi: 10.1016/j.radcr.2019.01.001

title: Breast involvement in mixed connective tissue disease.

authors

- Varma R
- Szilagyi S
- Harshan M

journal: Radiology case reports

journalAbbr: Radiol Case Rep

year: 2019

pubdate: 2019 Apr

volume: 14

issue: 4

pages: 430-435

abstract: To date, mammographic involvement in mixed connective tissue disease has not been described in the literature to the authors' knowledge. In addition, only 2 case reports have described panniculitis in mixed connective tissue disease. We present a patient with mixed connective tissue disease with coarse sheet-like calcifications on mammography bilaterally, which increased over time corresponding to rise in levels of anti-dsDNA. Biopsy showed fat necrosis with calcifications, chronic inflammation and fibrosis, vasculitis, as well as CD3, CD4, and CD8+ T cells and CD20+ B cells. These findings are similar histologically and mammographically to "lupus mastitis."

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6348285/

Record 933

pmcid: PMC6514336

pmid: 31110585

doi: 10.3889/oamjms.2019.303

title: Case Report: A Simple Thoraco - Abdominal Flap to Reconstruct Wide Defect of Radical Mastectomy in Squamous Cell Carcinoma of Breast.

authors

- Adiputra PAT
- Sudarsa IW

journal: Open access Macedonian journal of medical sciences

journalAbbr: Open Access Maced J Med Sci

year: 2019

pubdate: 2019 Apr 30

volume: 7

issue: 8

pages: 1356-1359

abstract: BACKGROUND: Squamous cell carcinoma (SCC) of the breast is very unusual. Thoraco-abdominal (TA) flap is a simple flap, and it is a rotation advancement fasciocutaneous flap. Here, we present a case of using TA flap for chest wall reconstruction in quick in-quick outpatient. CASE PRESENTATION: A Russian woman 48 years old presented enlarging lump on her left breast for the last 5 years. She was diagnosed as stage IV low-differentiated breast cancer luminal B and had a history of 4 cycles chemotherapy. Patient getting worsening and no response to chemotherapy. We decided to perform palliative radical mastectomy to improve quality of life. Primary skin closure was not possible due to the wide defect of skin and soft tissue. We decided to use TA flap to cover the defect. Histopathology result was compatible with SCC. CONCLUSION: TA flap can be the choice in patients with a wide defect of skin and soft tissue after a radical mastectomy. Given its simplicity and shorter operative time, TA flap is an ideal option for quick in-quick outpatient.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6514336/

Record 934

pmcid: PMC6794958

pmid: 31687020

doi: 10.1155/2019/6718761

title: Causes and Metabolic Consequences of Gynecomastia in Adult Patients.

authors

- Robeva R
- Elenkova A
- Zacharieva S

journal: International journal of endocrinology

journalAbbr: Int J Endocrinol

year: 2019

pubdate: 2019

volume: 2019

issue:

pages: 6718761

abstract: BACKGROUND: Gynecomastia (GM) is a benign enlargement of male breast due to glandular tissue proliferation. GM is a symptom of systemic or local hormonal disturbances, which could be associated with functional changes or pathological conditions. However, the long-lasting steroid imbalance in men with GM might exert negative influence on their metabolic health. METHODS: A total of 110 adult men with symptomatic GM were included in the present retrospective cross-sectional study. Anthropometric, metabolic, and hormonal data of the patients were collected. RESULTS: In almost 64% of GM patients, the underlying pathological condition was identified. Moreover, the development of GM was among the primary symptoms leading to the proper diagnosis in more than 40% of hypogonadal patients. The prevalence of metabolic syndrome (MS) was 53%; the highest prevalence of MS was found in patients with medication-induced GM and in the hypogonadal patients, whereas the lowest prevalence was observed in men with idiopathic postpubertal GM despite the similar degree of obesity. The lower testosterone levels were associated with more unfavorable lipid profile in the GM patients. CONCLUSION: The development of GM in adults might be an important symptom of an underlying gonadal disease. Moreover, it could be associated with an increased risk of metabolic disturbances. Our results support the need of detailed laboratory and hormonal investigations in patients with GM including targeted screening for metabolic disturbances. Further longitudinal studies are needed to evaluate the long-term consequences of sex hormones imbalance on cardiovascular morbidity and mortality in adults with GM.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6794958/

Record 935

pmcid: PMC6530271

pmid: 31143699

doi: 10.4103/ajm.AJM_184_18

title: Child and adolescent health in northwestern Syria: findings from Healthy-Syria 2017 study.

authors

- Terkawi AS
- Bakri B
- Alsadek AS

- Al-Hasan AH
- Alrahhal MS
- Alsaleh FM
- Alsatouf FA
- Arab MAI
- Jnaid H
- Hadid AA
- Terkawi RS
- Zahran MM
- Alghannam NA
- Altirkawi KA

journal: Avicenna journal of medicine

journalAbbr: Avicenna J Med

year: 2019

pubdate: 2019 Apr-Jun

volume: 9

issue: 2

pages: 61-74

abstract: OBJECTIVES: Since the uprising in 2011, there has been limited health-care data from inside Syria in the academic literature. This study aims to provide an updated account of pediatric health needs in the northwestern part of Syria; this should help inform the management and delivery of health-care services in this population. METHODS: This is a prospective study, using a data registry, of all pediatric patients seen in a single center in northwestern Syria, between February and December 2017. We used international classification of diseases (ICD-10) codes to define cases, and tested several covariates, including age, sex, season of the year, and conditions of living for possible correlations with major illness categories. RESULTS: We included 11,819 patients, of whom 5,288 (45%) were male and 6,531 (55%) were female. Collectively, these patients had 23,427 encounters. Respiratory diseases were the most encountered illnesses among all age groups (6320 [27%]), except late teen females, among whom gynecological/obstetric complaints dominated. Infectious diseases caused the greatest disease burden across all age groups, with upper respiratory tract infections (URTIs), infectious diarrhea, and otitis media representing almost half (47%) of all cases in this category. Nutritional deficiencies were diagnosed in 978 patients (8%), mostly in infants and toddlers (92%). We identified 1192 (17%) cases of acute diarrhea among all age groups, making it the second most common condition after URTIs. As compared to town residents, patients living in camps for internally displaced people accounted for more cases of infectious diarrhea (58%), chronic anemia (60%), and malnutrition (66%), especially severe acute malnutrition (76% of malnutrition cases). Vaccine-preventable illnesses represented a sizable category; we reported 69 cases of hepatitis A, 2 of poliomyelitis, 9 of pertussis, 37 of varicella, 11 of mumps, 8 of rubella, and 1 case of measles. CONCLUSION: We have identified urgent health-care issues in this population, including extreme malnutrition, high rates of infectious diseases, and high rates of teenage pregnancy. Also, we observed a relapse of some vaccine-preventable illnesses, such as mumps and rubella, which are likely associated with the decline in vaccination rates.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6530271/

Record 936

pmcid: PMC6594125

pmid: 31293322

doi: 10.4103/jhrs.JHRS_107_16

title: Chromosomal Aberrations in Primary Amenorrhea: A Retrospective Study.

authors

- Korgaonkar S
- Dhangar S
- Kulkarni V
- Kerketta L
- Vundinti BR

journal: Journal of human reproductive sciences

journalAbbr: J Hum Reprod Sci

year: 2019

pubdate: 2019 Apr-Jun

volume: 12

issue: 2

pages: 92-97

abstract: OBJECTIVES: The aim of this study was to estimate the frequency of chromosomal abnormalities and establish the association with clinical of factors such as secondary sexual characters and gonad development in primary amenorrhea (PA). STUDY DESIGN: The study was carried out in a large cohort of PA. The chromosomal aberrations were correlated with secondary sexual characters and anatomical abnormalities. MATERIALS AND METHODS: The data of 490 cases of PA were collected retrospectively. The chromosomal preparations were done from the peripheral blood and subjected to giemsa-trypsin-giemsa banding and karyotyped according to the International System of Human Cytogenetic Nomenclature 2013. The fluorescence in situ hybridization was carried out using centromeric and whole painting probes for X and Y chromosome. STATISTICAL ANALYSIS: Statistical analysis of the data was performed using online version of social science statistics software. RESULTS: A high frequency of abnormal uterus (81.9%) and ovaries (86.7%) were detected in our study. A total of 121 (24.7%) cases were identified with abnormal karyotype. The numerical chromosomal abnormalities were identified in 53 (43.8%) cases while structural abnormalities were identified in 32 (26.4%) cases. The XY karyotype was detected in 29.8% females with PA. The PA individuals with anatomical abnormalities (84.3%) had a high frequency (24.6%) of chromosomal aberrations. CONCLUSIONS: The present study concluded that cytogenetics plays an important role in precise diagnosis which helps in the management of PA. The cytogenetic analysis should be carried out to know the genetic basis of PA.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6594125/

Record 937

pmcid: PMC6878348

pmid: 31117335

doi: 10.4274/jcrpe.galenos.2019.2019.0027

title: Clinical and Biochemical Phenotype of Adolescent Males with Gynecomastia.

authors

- Lorek M
- Tobolska-Lorek D
- Kalina-Faska B
- Januszek-Trzciakowska A
- Gawlik A

journal: Journal of clinical research in pediatric endocrinology

journalAbbr: J Clin Res Pediatr Endocrinol

year: 2019

pubdate: 2019 Nov 22

volume: 11

issue: 4

pages: 388-394

abstract: OBJECTIVE: Gynecomastia is defined as a benign proliferation of male breast glandular tissue. Its prevalence during puberty varies between 50-60% and is also common in neonatal and elderly males. It develops mainly due to the disequilibrium between estrogen and androgen activity in breast tissue, where estradiol (E2) binds to estrogen receptors and stimulates ductal and glandular cells. The aim of this work was to investigate the relationship between sex hormone alterations and the natural history of gynecomastia. METHODS: Participants in this study were young males referred to an outpatient clinic, between January 2011 and February 2016, with breast enlargement. Thyroid function, liver function, hormone concentrations and tumor markers were measured and anthropometric assessment was conducted. RESULTS: Subjects comprised 93 males, aged 9 to 18 (mean±standard deviation age 13.8±2.6) years. In 63 of 93 (67.7%) the gynecomastia was confirmed and 28 were followed-up for a median period of three months. None of the boys showed any reduction in breast size during follow-up. There was no correlation between body mass index Z-score and breast size. Breast enlargement progressed in nine boys (32.1%). A positive correlation between estrogen to testosterone (E2/TTE) ratio and Tanner B stage (r=0.47; p=0.034) was observed. CONCLUSION: The E2/TTE ratio may be a helpful tool in diagnosing gynecomastia. Altered E2/TTE ratio might be responsible for a proportion of cases described previously as idiopathic. Additionally, weight loss does not imply reduction of breast size in boys. Nonetheless it should be the first step in the management of prolonged gynecomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6878348/

Record 938

pmcid: PMC6619787

pmid: 31312787

doi: 10.5152/ejbh.2019.4745

title: Clinical Evaluation of Breast in Childhood.

authors

- Karaayvaz S

journal: European journal of breast health

journalAbbr: Eur J Breast Health

year: 2019

pubdate: 2019 Jul

volume: 15

issue: 3

pages: 137-140

abstract: Childhood breast masses are mostly benign conditions starting from the newborn period continuing on to adolescence yet can cause high anxiety in the child and the family as well. As a complaint or physical finding, usually palpable mass, pain or discharge from the nipple is apparent in patients. All the clinicians interested in pediatric field should have full knowledge of immature and developing breasts so to proper diagnose and avoid overtreatment with unnecessary diagnostic or surgical procedures. Though malignancy or life-threatening disease has a very low probability during childhood, all child patients should be evaluated and followed up carefully. Especially training and then encouraging young people to periodically start self-assessment of the breasts after their 19th birthday while warning the ones who have had therapeutic chest radiation previously to begin self-assessment 8 years after the procedure or at 25 years of age whichever comes last, will be an appropriate intervention.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6619787/

Record 939

pmcid: PMC6629591

pmid: 31338317

doi: 10.5223/pghn.2019.22.4.407

title: Coexistence of Excessive Weight Gain and Celiac Disease in Children: An Unusual Familial Condition.

authors

- Calcaterra V
- Regalbuto C
- Madè A
- Magistrali M
- Leonard MM
- Cena H

journal: Pediatric gastroenterology, hepatology & nutrition

journalAbbr: Pediatr Gastroenterol Hepatol Nutr

year: 2019

pubdate: 2019 Jul

volume: 22

issue: 4

pages: 407-412

abstract: Excessive weight gain in children diagnosed with celiac disease (CD) is becoming more common. We describe 2 siblings (9-year and 6 months-old female and 6-year and 9 months-old male) with obesity showing attenuated gastrointestinal and atypical symptoms in which CD was diagnosed in the absence of a known family history of CD. After children's diagnosis, CD in their parents was also investigated. It was detected in their father affected by overweight. The presentation of patients with CD has changed. While patients with overweight and obesity commonly have symptoms such as abdominal pain, reflux, headache, and constipation due to lifestyle factors, CD should also be considered in patients with or without a family history of CD. Careful nutritional status assessment and follow-up monitoring after the diagnosis of CD are mandatory, especially in subjects who are already overweight at the presentation of this disease.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6629591/

Record 940

pmcid: PMC6754913

pmid: 31583153

doi: 10.1155/2019/5427207

title: Coincidental Central Precocious Puberty and Wilms Tumor in a 5-Year-Old Girl.

authors

- Kasongo L
- Forget P
- Nicolescu RC

journal: Case reports in pediatrics

journalAbbr: Case Rep Pediatr

year: 2019

pubdate: 2019

volume: 2019

issue:

pages: 5427207

abstract: Wilms tumor is the most frequent pediatric renal malignancy, and its usual presentation is an abdominal mass or hematuria. Unusual presentations have also been reported, such as paraneoplastic syndromes (acquired von Willebrand disease, sudden death due to pulmonary embolism, and Cushing syndrome). These conditions can precede, occur concomitantly, or present in a later phase of tumor development. Precocious puberty, as paraneoplastic endocrine syndrome, has already been described in children with malignant tumors (brain, gonadal, adrenal tumors, and hepatoblastoma). However, little is known about central precocious puberty, as paraneoplastic manifestation of nephroblastoma or secondary to its specific chemotherapy. Here, we report a case of Wilms tumor and simultaneous precocious puberty in a 5-year-old girl. The initial diagnosis was premature telarche, but the clinical and biological pubertal progression changed our diagnosis to idiopathic central precocious puberty. Chemotherapy and nephrectomy were well tolerated, and we began treatment with a gonadotropin-releasing hormone agonist which showed favorable outcomes over the short term. We highlight the need for early diagnosis and work-up in all patients of precocious puberty, in order to institute timely management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6754913/

Record 941

pmcid: PMC7061682

pmid: 32158861

doi: 10.1016/j.jpra.2018.12.008

title: Correction of the tuberous breast deformity in a prepubescent male patient: A surgical approach to an unusual problem.

authors

- Gorvetzian J
- Funderburk C
- Copeland-Halperin LR
- Nigriny J

journal: JPRAS open

journalAbbr: JPRAS Open

year: 2019

pubdate: 2019 Mar

volume: 19

issue:

pages: 98-105

abstract: PURPOSE: The management of the tuberous breast deformity in the female patient is well described. However, the presence of this variant in male patients is particularly rare, and few reports on the management of this condition are available. CASE PRESENTATION: A 12-year-old prepubescent male with bilateral gynecomastia and tuberous breast deformities was referred to our department for treatment. Our surgical management, including free nipple areolar complex harvest, mastectomy, removal of excess skin and subsequent nipple grafting, is presented in detail. We observed a cosmetically acceptable result with restoration of a masculine-appearing nipple-areolar complex and good patient satisfaction at 6-month follow-up. CONCLUSIONS: Tuberous breast deformities in male patients are rare. Our treatment of a prepubertal male patient with this deformity using mastectomies and free nipple areolar complex grafting provided a cosmetically

acceptable result. Here, we review the current literature on tuberous breast deformities in males and describe our approach to treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7061682/

Record 942

pmcid: PMC6374696

pmid: 30809582

doi: 10.1016/j.ijwd.2018.08.005

title: Cutaneous complications associated with breast augmentation: A review.

authors

- Chopra S
- Marucci D

journal: International journal of women's dermatology

journalAbbr: Int J Womens Dermatol

year: 2019

pubdate: 2019 Feb

volume: 5

issue: 1

pages: 73-77

abstract: Breast augmentation is one of the most popular and safe cosmetic procedures performed by plastic surgeons worldwide. Although breast implants are available in a number of different materials, silicone-filled implants remain the most common type. However, prior to the development of breast implants, various materials were injected into the soft tissues of the breasts to increase breast volume, which caused cutaneous complications and disfigurement. This review details the history of breast augmentation, the current methods used in augmentation surgery, and associated cutaneous complications.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6374696/

Record 943

pmcid: PMC7288886

pmid: 32537301

doi: 10.1097/GOX.0000000000002554

title: Death by Implants: Critical Analysis of the FDA-MAUDE Database on Breast Implant-related Mortality.

authors

- Abi-Rafeh J
- Safran T
- Al-Halabi B
- Dionisopolous T

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2019

pubdate: 2019 Dec

volume: 7

issue: 12

pages: e2554

abstract: UNLABELLED: Since the 1992 moratorium by the Food and Drug Administration (FDA), the debate on the association of breast implants with systemic illnesses has been ongoing. Breast implant-associated anaplastic large cell lymphoma has also raised significant safety concerns in recent years. METHODS: A systematic search of the Manufacturer and User Facility Device Experience (MAUDE) database was performed to identify all cases of breast implant-associated deaths reported to the FDA. RESULTS: The search identified 50 reported cases of apparent implant-related mortality; breast implant-associated anaplastic large cell lymphoma comprised the majority of fatal outcomes (n = 21, 42%), followed by lymphoma (n = 4, 8%), breast cancer (n = 3, 6%), pancreatic cancer (n = 2, 4%), implant rupture (n = 2, 4%), and postoperative infections (n = 2, 4%). Single cases (n = 1, 2% each) of leukemia, small bowel cancer, lung disease, pneumonia, autoimmune and joint disease, amyotrophic lateral sclerosis, liver failure, and sudden death, and 2 cases (4%) of newborn deaths, to mothers with breast implants, were also identified. A literature review demonstrated that 54% of alleged implant-related deaths were not truly associated with breast implant use: the majority of these reports (82%) originated from the public and third-party sources, rather than evidence-based reports by health-care professionals and journal articles. CONCLUSIONS: Although there exists a need for more comprehensive reporting in federal databases, the information available should be considered for a more complete understanding of implant-associated adverse outcomes. With only 46% of FDA-reported implant-related deaths demonstrated to be truly associated with breast implant use, there exists a need for public awareness and education on breast implant safety.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7288886/

Record 944

pmcid: PMC6497076

pmid: 31027550

doi: 10.1016/j.ecl.2019.01.005

title: Dermatologic Conditions in Transgender Populations.

authors

- Yeung H
- Kahn B
- Ly BC
- Tangpricha V

journal: Endocrinology and metabolism clinics of North America

journalAbbr: Endocrinol Metab Clin North Am

year: 2019

pubdate: 2019 Jun

volume: 48

issue: 2

pages: 429-440

abstract: Transgender persons receiving gender-affirming hormone therapy and procedures may face specific skin conditions. Skin diseases in transgender patients often are underdiagnosed and underrecognized despite their important impact on quality of life and mental health. This article discusses pathophysiology, diagnosis, and treatment of common skin diseases in the transgender populations. For transmasculine patients, conditions include acne vulgaris and male pattern hair loss. For transfeminine patients, conditions include hirsutism, pseudofolliculitis barbae, and melasma. Postprocedural keloids and other cutaneous complications are discussed. Unique aspects of skin health in transgender persons should be considered in the context of multidisciplinary gender-affirming care.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6497076/

Record 945

pmcid: PMC6502167

pmid: 30889150

doi: 10.23750/abm.v90i1.6607

title: Diagnostic techniques and multidisciplinary approach in idiopathic granulomatous mastitis: a revision of the literature.

authors

- Maione C
- Palumbo VD
- Maffongelli A
- Damiano G
- Buscemi S
- Spinelli G
- Fazzotta S
- Gulotta E
- Buscemi G
- Lo Monte AI

journal: Acta bio-medica : Atenei Parmensis

journalAbbr: Acta Biomed

year: 2019

pubdate: 2019 Jan 23

volume: 90

issue: 1

pages: 11-15

abstract: Idiopathic granulomatous mastitis (IGM) is a chronic benign inflammatory disease of the breast that may mimic breast cancer. It is most common in parous young fertile women, although it can occur in nulliparous women and in men. IGM is an idiopathic disease due to the influence of some environmental factors in genetically predisposed subjects. Several pathogenic hypothesis have been proposed in the last years (autoimmune, hormonal, infective genesis). IGM presents as a painful palpable mass located in one of the two udders. The skin is usually normal but could present signs of inflammation with or without lymph nodes involvement. Ultrasonography, mammography, magnetic resonance can be diagnosed an IGM, but pathognomonic radiological signs has not yet reported in literature. Biopsy findings show granulomatous lesion centered on the breast lobule, as in granulomatous mastitis induced by tuberculosis or sarcoidosis. The aim of this review of literature is to verify the development of new advanced diagnostic techniques and multidisciplinary approach for this condition. In the last years innovative approaches have modified IGM diagnosis and therapy, avoiding surgery in most of cases, introducing a more conservative medical approach based on recent etiopathological hypothesis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6502167/

Record 946

pmcid: PMC6319696

pmid: 30566415

doi: 10.1097/JCP.0000000000000979

title: Effect of Brexpiprazole on Prolactin: An Analysis of Short- and Long-Term Studies in Schizophrenia.

authors

- Ivkovic J
- Lindsten A
- George V
- Eriksson H
- Hobart M

journal: Journal of clinical psychopharmacology

journalAbbr: J Clin Psychopharmacol

year: 2019

pubdate: 2019 Jan/Feb

volume: 39

issue: 1

pages: 13-19

abstract: BACKGROUND: Hyperprolactinemia is an undesirable effect of most antipsychotics because of D2-receptor blockade. We assessed the effect of the D2-receptor partial agonist brexpiprazole on prolactin, based on pooled data from three 6-week, randomized, placebo-controlled studies and two open-label extension studies in patients with schizophrenia. METHODS: In the short-term studies, patients received 0.25, 1, 2, 4 mg brexpiprazole or placebo; or flexible-dose brexpiprazole (2-4 mg/d), placebo, or active reference. The extension studies were 52-week, flexible-dose (1-4 mg/d) studies. We studied changes from baseline and shifts in prolactin status in patients with normal or elevated prolactin levels at baseline, and prolactin-related treatment-emergent adverse events (TEAEs). RESULTS: Median changes from baseline to week 6 in brexpiprazole-treated patients in short-term studies were as follows: 3.63 ng/mL (females), 0.26 ng/mL (males); placebo: -2.15 ng/mL (females), -1.08 ng/mL (males).Median changes from baseline to week 52 in long-term studies were 0.60 ng/mL (females) and 0.18 ng/mL (males). Prolactin levels in patients with baseline values greater than 1× upper limit of normal tended to decrease over time regardless of previous treatment.The proportions of brexpiprazole-treated patients with greater than 3× upper limit of normal postbaseline prolactin values in short-term studies were as follows: 1.5% (females), 1.6% (males); placebo: 3.6% (females), 3.4% (males). Corresponding figures in long-term studies were 5.3% (females) and 2.0% (males).In short-term studies, the incidence of prolactin-related TEAEs was 1.8% for brexpiprazole and 0.6% for placebo. In long-term studies, the incidence of prolactin-related TEAEs was 1.7%. CONCLUSIONS: Small changes in prolactin levels, low proportions of patients with postbaseline elevated prolactin values, and low incidence of prolactin-related TEAEs were observed after treatment with brexpiprazole.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6319696/

Record 947

pmcid: PMC6348527

pmid: 30719081

doi: 10.1177/1756287218820807

title: Efficacy and safety of fexapotide triflutate in outpatient medical treatment of male lower urinary tract symptoms associated with benign prostatic hyperplasia.

authors

- Shore N
- Tutrone R
- Roehrborn CG

journal: Therapeutic advances in urology

journalAbbr: Ther Adv Urol

year: 2019

pubdate: 2019 Jan-Dec

volume: 11

issue:

pages: 1756287218820807

abstract: Male lower urinary tract symptoms (LUTS) is an increasingly important problem for the majority of late middle aged and elderly men. Fexapotide triflutate (FT) is a first in-class compound given by local injection via the transrectal intraprostatic route under ultrasound guidance. Data from >1700 FT and control injections in prospective randomized blinded controlled multicenter trials are reviewed and discussed in relation to current developments in the field of treatments for LUTS associated with benign prostatic hyperplasia (BPH). Long-term studies of FT in the United States have shown statistically significant improvement in BPH symptoms and objective outcomes including significant reduction in both spontaneous acute urinary retention as well as the subsequent incidence of BPH surgery. FT has been shown to be well tolerated with an excellent safety profile, and is an efficacious clinic-based treatment for BPH involving an intraprostatic injection that requires only a few minutes to administer, with no catheter nor anesthesia requirements.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6348527/

Record 948

pmcid: PMC6553508

pmid: 30694917

doi: 10.1097/GME.0000000000001292

title: Efficacy and safety of ospemifene in postmenopausal women with moderate-to-severe vaginal dryness: a phase 3, randomized, double-blind, placebo-controlled, multicenter trial.

authors

- Archer DF
- Goldstein SR
- Simon JA
- Waldbaum AS
- Sussman SA
- Altomare C
- Zhu J
- Yoshida Y
- Schaffer S
- Soulban G

journal: Menopause (New York, N.Y.)

journalAbbr: Menopause

year: 2019

pubdate: 2019 Jan 28

volume: 26

issue: 6

pages: 611-621

abstract: OBJECTIVE: To evaluate the safety and efficacy of ospemifene for the treatment of moderate to severe vaginal dryness in postmenopausal women with vulvovaginal atrophy (VVA). METHODS: This 12-week,

multicenter, double-blind phase 3 study randomized postmenopausal women (aged 40-80 years) with VVA and moderate to severe vaginal dryness as their most bothersome symptom to daily oral ospemifene 60 mg or placebo. Coprimary efficacy endpoints included changes from baseline to week 12 in percentages of vaginal parabasal and superficial cells, vaginal pH, and vaginal dryness severity with ospemifene versus placebo; other secondary endpoints were evaluated (weeks 4, 8, and 12). Safety was assessed by treatment-emergent adverse events (TEAEs) and endometrial biopsies. RESULTS: Women (n = 631; ospemifene [n = 316], placebo [n = 315]) had a mean age of 59.8 years, a mean body mass index of 27.2 kg/m, and most were white. Ospemifene significantly improved (P < 0.0001) the percentages of parabasal and superficial cells, vaginal pH, and severity of vaginal dryness severity compared with placebo at week 12; significant between-group differences were noted by week 4. Secondary endpoints of dyspareunia (P < 0.001), maturation value (P < 0.0001), and the Female Sexual Function Index (P < 0.05) also significantly improved with ospemifene versus placebo at week 12. Significantly more women responded (31.5% vs 6.0%; P < 0.0001) or were satisfied (49.2% vs 33.8%; P = 0.0007) with ospemifene versus placebo at week 12. No unexpected TEAEs, treatment-related serious TEAEs, thrombotic events, or endometrial hyperplasia or carcinoma were observed. CONCLUSIONS: Ospemifene was effective and well tolerated for the treatment of moderate-to-severe vaginal dryness in postmenopausal women with VVA.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6553508/

Record 949

pmcid: PMC6784621

pmid: 31620613

doi: 10.12701/yujm.2019.00017

title: Estrogen-secreting adrenocortical carcinoma.

authors

- Jeong Y
- Cho SC
- Cho HJ
- Song JS
- Kong JS
- Park JW
- Ku YH

journal: Yeungnam University journal of medicine

journalAbbr: Yeungnam Univ J Med

year: 2019

pubdate: 2019 Jan

volume: 36

issue: 1

pages: 54-58

abstract: Adrenocortical carcinoma is a rare type of endocrine malignancy with an annual incidence of approximately 1-2 cases per million. The majority of these tumors secrete cortisol, and a few secrete aldosterone or androgen. Estrogen-secreting adrenocortical carcinomas are extremely rare, irrespective of the secretion status of other adrenocortical hormones. Here, we report the case of a 53-year-old man with a cortisol and estrogen-secreting adrenocortical carcinoma. The patient presented with gynecomastia and abdominal discomfort. Radiological assessment revealed a tumor measuring 21×15.3×12 cm localized to the retroperitoneum. A hormonal evaluation revealed increased levels of estradiol, dehydroepiandrosterone sulfate, and cortisol. The patient underwent a right adrenalectomy, and the pathological examination revealed an adrenocortical carcinoma with a Weiss' score of 6. After surgery, he was treated with adjuvant radiotherapy.

Twenty-one months after treatment, the patient remains alive with no evidence of recurrence.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6784621/

Record 950

pmcid: PMC6797486

pmid: 31632211

doi: 10.1055/s-0039-1696998

title: Finding Consensus After Two Decades of Breast Implant-Associated Anaplastic Large Cell Lymphoma.

authors

- Clemens MW
- DeCoster RC
- Fairchild B
- Bessonov AA
- Santanelli di Pompeo F

journal: Seminars in plastic surgery

journalAbbr: Semin Plast Surg

year: 2019

pubdate: 2019 Nov

volume: 33

issue: 4

pages: 270-278

abstract: Breast implant-associated anaplastic large cell lymphoma (BIA-ALCL) is an emerging and indolent, but potentially fatal cancer of the immune system that can develop around textured-surface breast implants. The World Health Organization first recognized BIA-ALCL as a unique clinical entity in 2016. To date, over 600 confirmed cases have been reported worldwide. BIA-ALCL most commonly presents with disease confined to the capsule, as a seroma or a mass adjacent to the implant. While BIA-ALCL has a fairly indolent clinical course, with an excellent prognosis in early stage disease, disseminated cancer and death have also been reported. In this review, the authors focus on the early diagnosis and treatment, including reconstructing the breast following BIA-ALCL, and also discuss recently updated National Comprehensive Cancer Network guidelines. They also review the current epidemiology and risk factors associated with BIA-ALCL. Finally, they discuss important medicolegal considerations and the bioethics surrounding the continued use of textured-surface breast implants.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6797486/

Record 951

pmcid: PMC6307343

pmid: 30603606

doi: 10.1016/j.rmcr.2018.12.001

title: Galactorrhea, mastodynia and gynecomastia as the first manifestation of lung adenocarcinoma. A case report.

authors

- Lazopoulos A
- Krimiotis D
- Schizas NC

- Rallis T
- Gogakos AS
- Chatzinikolaou F
- Tsiouda T
- Zarogoulidis P
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- Paliouras D
- Barbetakis N

journal: Respiratory medicine case reports

journalAbbr: Respir Med Case Rep

year: 2019

pubdate: 2019

volume: 26

issue:

pages: 146-149

abstract: Gynecomastia with mastodynia and galactorrhea as a paraneoplastic syndrome due to lung cancer with complete response after surgical excision is rare. A 62-year-old Caucasian male presented with mastodynia, galactorrhea and right breast enlargement. Chest x-ray revealed a left upper lobe tumor. The patient had high levels of serum beta-human chorionic gonadotropin (b-HCG) and prolactine. Complete staging was negative for metastases. A typical left upper lobectomy with radical mediastinal lymph node dissection was performed. Pathology report was consistent with a poorly differentiated adenocarcinoma (T 2 N 1 M 0). Immunohistochemically, multinucleate cells and occasional mononucleate tumor cells showed positivity for human chorionic gonadotropin. The patient received adjuvant chemotherapy with cisplatin - navelbine. One year later physical examination showed regression of both gynecomastia and mastodynia and there was no nipple discharge, while he is free from local or distant metastatic disease and the b-HCG level is normal (1,59 mIU/ml). This case represents a very rare, first manifestation of lung cancer. Galactorrhea, mastodynia and gynecomastia were the initial symptoms, which totally resolved following the successful surgical resection and adjuvant chemotherapy. In this case, prolactin and b-HCG are useful biomarkers during follow up for checking local or distal recurrence of the disease.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6307343/

Record 952

pmcid: PMC6431701

pmid: 30956501

doi: 10.1007/s13224-018-1184-7

title: Gestational Gigantomastia in the Setting of Myasthenia Gravis.

authors

- Mangla M
- Chhatwal J
- Nautiyal R
- Prasad D

journal: Journal of obstetrics and gynaecology of India

journalAbbr: J Obstet Gynaecol India

year: 2019

pubdate: 2019 Apr

volume: 69

issue: Suppl 1

pages: 84-87

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6431701/

Record 953

pmcid: PMC6487774

pmid: 31065379

doi: 10.1177/2055102919844492

title: Giving voice to my childbirth experiences and making peace with the birth event: the effects of the first childbirth on the second pregnancy and childbirth.

authors

- Rania N

journal: Health psychology open

journalAbbr: Health Psychol Open

year: 2019

pubdate: 2019 Jan-Jun

volume: 6

issue: 1

pages: 2055102919844492

abstract: This autoethnographic study describes the author's waterbirthing experience to evidence the relationship between fear of childbirth and communication with, and support from, healthcare professionals and the medical process during labour and delivery. The study provides a rereading of the author's experience, which demonstrates how the traumatic consequences of a first childbirth influenced the experience of a second pregnancy and childbirth. This account indicates how lack of training and inadequate communication by medical staff can lead to traumatic childbirth experience. The study enhances understanding of womens' transition to motherhood with implications for practice, education and research of midwives and other medical providers.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6487774/

Record 954

pmcid: PMC6818064

pmid: 31508989

doi: 10.1308/rcsann.2019.0108

title: Gynaecomastia: a cosmetic concern or a concerning clinical sign?

authors

- Whitehead IJ
- Fernandes L
- Haris O
- Chagla L

journal: Annals of the Royal College of Surgeons of England

journalAbbr: Ann R Coll Surg Engl

year: 2019

pubdate: 2019 Nov

volume: 101

issue: 8

pages: e184-e186

abstract: Gynaecomastia is common and often physiological, leading to frequent dismissal as a cosmetic condition. It can however be a symptom of underlying hormone imbalance, which requires further assessment to exclude underlying sinister pathology. We discuss a rare cause of gynaecomastia that demonstrates the importance of a holistic approach to patient assessment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6818064/

Record 955

pmcid: PMC6693125

pmid: 31409277

doi: 10.1186/s12879-019-4332-5

title: Gynecomastia in HIV-positive adult men receiving efavirenz-based antiretroviral therapy at Newlands clinic, Harare, Zimbabwe.

authors

- Shawarira-Bote S
- Shamu T
- Chimbetete C

journal: BMC infectious diseases

journalAbbr: BMC Infect Dis

year: 2019

pubdate: 2019 Aug 13

volume: 19

issue: 1

pages: 715

abstract: BACKGROUND: Gynecomastia is known to occur in some men taking an efavirenz-based antiretroviral therapy (ART) regimen. However, the incidence and outcomes of gynecomastia are not known in Zimbabwe. We described the characteristics and outcomes of gynecomastia among male patients on an efavirenz-based ART regimen. METHODS: We conducted a retrospective cohort review of data of all male patients aged ≥18 years taking an efavirenz-based regimen at Newlands Clinic, Harare, Zimbabwe before 31 March 2017. The primary outcome was gynecomastia as defined by breast/nipple enlargement reported by patient and confirmed by clinical palpation. Routinely collected data on demographics, baseline CD4, body mass index, duration on efavirenz, clinical presentation and outcomes were extracted from the clinic database and analysed using STATA 12.1. We investigated for any associations with concomitant medicines using cox regression. RESULTS: We analysed data for 1432 men with a median age of 40 years (IQR: 33-48). Half of the patients were in WHO stage 1 at ART commencement. Median body mass index and CD4 count at efavirenz commencement was 21 (IQR: 19-23) and 260 cells/mm 3 (IQR: 126-412) respectively. The incidence of gynecomastia was 22/1000 person-years (IQR: 17.3-27.8). Over half of the cases (58%) were bilateral and 75% of all cases developed within two years of starting efavirenz. There were no significant associations with concomitant use of isoniazid (HR: 0.95, p = 0.87) or amlodipine (HR: 0.43, p = 0.24). Gynecomastia resolved in 83.5% of cases following withdrawal of efavirenz with a median time to resolution of 3 months (IQR: 2-9). CONCLUSION: The incidence of gynecomastia among patients taking efavirenz-based ART was low with most

cases developing early on during treatment. Most cases resolved completely after withdrawing efavirenz.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6693125/

Record 956

pmcid: PMC7040313

pmid: 32127876

doi: 10.4314/ahs.v19i4.22

title: How do reproductive age women perceive breast cancer screening in Ethiopia? A qualitative study.

authors

- Agide FD
- Garmaroudi G
- Sadeghi R
- Shakibazadeh E
- Yaseri M
- Koricha ZB

journal: African health sciences

journalAbbr: Afr Health Sci

year: 2019

pubdate: 2019 Dec

volume: 19

issue: 4

pages: 3009-3017

abstract: BACKGROUND: Breast cancer remains one of the deadliest non-communicable diseases in the world. In Ethiopia, breast cancer accounts for 33.4% of total cancer diagnosis in women. OBJECTIVE: This study aims to explore perception about breast screening behavior among reproductive age women. METHODS: This qualitative study was conducted as a baseline to identify gaps to design interventions that will enhance breast screening uptake among reproductive age women. Six focus group discussions and 9 in-depth interviews were conducted with women and health workers respectively. Semi-structured questions were used. Data analysis was analyzed by Atlas.ti. 7 and the ideas were put in direct quotation and narration. RESULTS: Lack of awareness is the preceding problem for self-susceptibility to breast cancer as well as for having breast screening. Majority of women thought that the cause of breast cancer was a sin (supernatural power). Self-efficacy and cues to action were the most important correlates of the perception owing to fear of socio-cultural stigma and discrimination. CONCLUSION: All health belief model constructs identified a critical problem for adaptation of behavior. Therefore, this gives the opportunity to design and develop community-based intervention and explore new intervention mechanism with an accurate method.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7040313/

Record 957

pmcid: PMC6880085

pmid: 32431424

doi: 10.1177/0024363919874606

title: Humility before New Scientific Evidence: We No Longer Have Moral Certainty that "Brain Death" Is True Death.

authors

- Alexander I

journal: The Linacre quarterly

journalAbbr: Linacre Q

year: 2019

pubdate: 2019 Nov

volume: 86

issue: 4

pages: 314-326

abstract: UNLABELLED: In light of the current medical evidence, physicians and ethicists no longer have the moral certainty that Pope John Paul II deemed vital for vital organ transplantation. The current medical evidence suggests that some brain-dead patients do seem to exhibit signs of global integration. This article will analyze the interpretations of this evidence, both from ethicists for and against the neurological criteria. Even the strongest arguments in favor of the neurological criteria in light of the new evidence fail to prove that the patient with total brain failure (TBF) has died. Furthermore, the recent Jahi McMath case demonstrates a new and alarming reality that there is no way in practice to distinguish global ischemic penumbra from TBF. In the absence of the moral certainty required by Pope John Paul II, it is clear that the Catholic Church should immediately call for a halt on vital organ transplantation. SUMMARY: This article analyzes the current medical evidence about brain death and argues that we have lost the moral certainty that Pope John Paul II deemed necessary for vital organ transplantation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6880085/

Record 958

pmcid: PMC6352236

pmid: 30621064

doi: 10.3390/jcm8010056

title: Imperforate Hymen: A Comprehensive Systematic Review.

authors

- Lee KH
- Hong JS
- Jung HJ
- Jeong HK
- Moon SJ
- Park WH
- Jeong YM
- Song SW
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- Son MJ
- Lim JJ
- Shin JI

journal: Journal of clinical medicine

journalAbbr: J Clin Med

year: 2019

pubdate: 2019 Jan 7

volume: 8

issue: 1

pages:

abstract: Imperforate hymen (IH) is an uncommon congenital anomaly of the female genital tract, with the hymen completely obstructing the vaginal opening. Despite the simple diagnosis and treatment of IH, missed or delayed diagnosis is often a clinical problem owing to its low incidence, nonspecific symptoms, or insufficient physical examination. The aim of this study is to identify the characteristics, clinical presentations, treatment modalities, and outcomes of imperforate hymen patients. In this study, a literature search of PubMed, Scopus and Medline databases was performed for sources published up to 3 July 2018 for English-language studies with the term "imperforate hymen". The literature review identified 251 citations and 155 articles (143 case reports, 12 case series) containing 253 patients who were finally included (two papers were not written in English). Among 236 postnatal patients, the mean age of the patients was 10.7 ± 4.7 years. Abdominal pain (54.2%), urinary retention (20.3%), abnormal menstruation (14.0%), dysuria (9.7%), increased urinary frequency (5.1%), severe presentation of renal failure (n = 5, 2.1%), and urinary tract infection (n = 1, 0.4%) were presented. Most patients diagnosed with the condition underwent surgical treatment (83.5%), most of whom were treated via a hymenotomy (35.2%) and hymenectomy (36.4%), and the use of prophylactic antibiotics were only used in 7 patients. There were no differences in outcomes between two surgical methods. In addition, 141 (59.7%) patients showed improvement and 5 deceased patients were not related to IH or the operation itself; Complications, such as vaginal adhesion, were only noted in 6.6% of patients. In addition, among 17 cases of newborns with a diagnosis of IH before birth, hymenectomy (n = 5, 29.4%) and hymenotomy (n = 9, 52.9%) were the main treatment modalities and showed improved prognosis in 52.9% of newborns. Because IH diagnosis is easy and postsurgical prognosis is good, clinicians should carefully examine every female patient at birth. IH should be considered regarding adolescent girls with abdominal pain, lower back pain, or urinary retention, and perform appropriate physical examinations of the genital introitus. In addition, accurate diagnosis as IH, not misdiagnosing as vaginal septum or agenesis, is important to prevent severe complications such as stricture and ascending infection.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6352236/

Record 959

pmcid: PMC6797495

pmid: 31632208

doi: 10.1055/s-0039-1696988

title: Implant Reconstruction in Nipple Sparing Mastectomy.

authors

- Chu CK
- Davis MJ
- Abu-Ghname A
- Winocour SJ
- Losken A
- Carlson GW

journal: Seminars in plastic surgery

journalAbbr: Semin Plast Surg

year: 2019

pubdate: 2019 Nov

volume: 33

issue: 4

pages: 247-257

abstract: Nipple sparing mastectomy has been popularized in the modern era of breast cancer treatment due to

its touted advantages with regard to resultant body image and reconstructive outcome. Implant-based techniques remain the most prevalent means of breast reconstruction. Special considerations regarding patient selection and technique are reviewed for implant reconstruction in the setting of mastectomy with nipple preservation. Applications for prepectoral and direct-to-implant reconstruction are discussed and published outcomes are summarized.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6797495/

Record 960

pmcid: PMC6390125

pmid: 30760630

doi: 10.1042/BSR20181776

title: Increase of glandular epithelial cell clusters by an external volume expansion device promotes adipose tissue regeneration by recruiting macrophages.

authors

- Chen X
- He Y
- Xu A
- Deng Z
- Feng J
- Lu F
- Yuan Y

journal: Bioscience reports

journalAbbr: Biosci Rep

year: 2019

pubdate: 2019 Feb 28

volume: 39

issue: 2

pages:

abstract: Background: There is a clinical need for the use of engineered adipose tissue in place of surgical reconstruction. We previously found that the external volume expansion (EVE) device increased special cell clusters in well-vascularized connective stroma during adipose regeneration. However, the origin of these cell clusters and their role in adipose tissue regeneration remain unknown. Aim: In the present study, we evaluated EVE in the construction of expanded prefabricated adipose tissue (EPAT) in a rat model. Methods: Rats were randomized into an EVE suction group and a control group, with 24 rats in each group. The structure and origin of the special cell clusters were determined by hematoxylin and eosin staining, and immunohistochemistry; their role in adipose tissue regeneration was investigated by immunohistochemistry and Western blot analyses. Results: Special cell clusters began to increase at week 1 with a peak at week 4, and then receded from weeks 8 to 12. Clusters were identified as glandular epithelial cells as determined by their gland-like structure and expression of specific markers. The cell clusters induced significant infiltration of macrophage antigen-2 (Mac-2) positive macrophages by secreting monocyte chemoattractant protein-1 (MCP-1) at the early stage of suction. Subsequently, these infiltrated macrophages expressed massive vascular endothelial growth factor (VEGF) to promoted angiogenesis. Conclusion: EVE generated glandular epithelial cell clusters, which recruited macrophages to promote angiogenesis and subsequent adipose tissue regeneration. These findings shed light on the mechanisms underlying the effects of EVE devices on adipose tissue regeneration.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6390125/

Record 961

pmcid: PMC6327217

pmid: 30680198

doi: 10.3892/mco.2018.1773

title: Inflammatory breast cancer in 210 patients: A retrospective study on epidemiological, anatomo-clinical features and therapeutic results.

authors

- Manai M
- Finetti P
- Mejri N
- Athimni S
- Birnbaum D
- Bertucci F
- Rahal K
- Gamoudi A
- Chaffanet M
- Manai M
- Boussen H

journal: Molecular and clinical oncology

journalAbbr: Mol Clin Oncol

year: 2019

pubdate: 2019 Feb

volume: 10

issue: 2

pages: 223-230

abstract: To report epidemiological and anatomo-clinical features within a retrospective series of inflammatory breast cancer and to evaluate prognostic factors. This retrospective study included 210 Tunisian patients presenting a clinically diagnosed IBC, treated at the Institute Salah Azaiez (ISA) of Tunis, Tunisia, from 2008 to 2013. We collected data on epidemiology, anatomo-clinical and biological features and histologic response to neoadjuvant therapy. Overall and disease-free survivals were calculated by Kaplan-Meier method and compared by log-rank tests and Cox's models were used to identify prognostic factors impacting survival. The 210 IBC patients had a median age of 42 years (24-62) and 15% of them were aged less than 35 years. Mean age at menarche was 13 years and 45% had their 1st childbirth before 23 years. On histology, grades III represented 42% of cases, hormone receptors were negative in 59%, HER2 over-expressed in 32, 25% of our IBC cases had a triple negative profile and Ki-67 was >20% in 53% of cases. High pathological grade III was significantly correlated to TN subtype (58%) (Fisher's exact test, P=7.5×10 -3). Further, high Ki-67 expression (>20%) was evident in the TN subtype (84%) (Fisher's exact test, P=3.7×10 -4). After neoadjuvant therapy (and trastuzumab in 88 and 69% of HER2+ patients, respectively), we observed 49% of objective clinical responses and 35% of pathological complete response (pCR) and >3 axillary lymph nodes were invaded on a resected tumor in 55% of cases. Overall survival (OS) was associated with age at menarche (Wald-test, P=2.2×10 -2) and metastases at diagnosis (Wald-test, P=2.4×10 -2). Reaching a pCR was correlated with a better metastasis-free survival (MFS), (Fisher's exact test, P=3.6×10 -2).

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6327217/

Record 962

pmcid: PMC6764051

pmid: 31552775

doi: 10.1177/1557988319871423

title: Influence of Postoperative Finasteride Therapy on Recurrence of Gynecomastia After Mastectomy in Men Taking Finasteride for Alopecia.

authors

- Lee SG
- Park PJ
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- Koo BH
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journal: American journal of men's health

journalAbbr: Am J Mens Health

year: 2019

pubdate: 2019 Sep-Oct

volume: 13

issue: 5

pages: 1557988319871423

abstract: Finasteride is commonly used for treatment of alopecia. Because finasteride is a cause of gynecomastia, there is concern regarding the continuation of finasteride therapy after mastectomy. No studies have been performed to determine whether finasteride should be continued after mastectomy when gynecomastia occurs in patients taking finasteride for the treatment of alopecia. The researchers studied the effects of finasteride on gynecomastia recurrence after mastectomy in men with gynecomastia taking finasteride for alopecia. The researchers retrospectively evaluated 1,673 patients with gynecomastia who underwent subcutaneous mastectomy with liposuction at Damsoyu Hospital from January 2014 to December 2016. In total, 52 of the patients were taking finasteride for alopecia before surgery and continued to use it in the same manner after mastectomy. Ultrasonography was performed 1 year after mastectomy. The patients' median age was 26.5 (24.75-30) years. All 52 patients had bilateral gynecomastia. The median duration of finasteride therapy before and after surgery was 12 (5-25.75) and 33 (27.5-40.5) months, respectively. There were no statistically significant differences between the groups with and without the use of finasteride in relation to postoperative complications and recurrence rates. Taking finasteride seems to have little effect on recurrence in patients with alopecia who have undergone surgical treatment of gynecomastia. Surgeons may recommend continuous finasteride therapy in patients with alopecia who wish to take finasteride after mastectomy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6764051/

Record 963

pmcid: PMC6501645

pmid: 30939983

doi: 10.1098/rsob.190006

title: Is breast implant-associated anaplastic large cell lymphoma a hazard of breast implant surgery?

authors

- Fitzal F

- Turner SD
- Kenner L

journal: Open biology

journalAbbr: Open Biol

year: 2019

pubdate: 2019 Apr 26

volume: 9

issue: 4

pages: 190006

abstract: Breast implant-associated anaplastic large cell lymphoma (BIA-ALCL) may occur after reconstructive or aesthetic breast surgery. Worldwide, approximately 1.7 million breast implant surgeries are performed each year. To date, over 500 cases of BIA-ALCL have been reported around the world, with 16 women having died. This review highlights the most important facts surrounding BIA-ALCL. There is no consensus regarding the true incidence rate of BIA-ALCL as it varies between countries, is probably significantly under-reported and is difficult to estimate due to the true number of breast prostheses used largely being unknown. BIA-ALCL develops in the breast mostly as a seroma surrounding the implant, but contained within the fibrous capsule, or more rarely as a solid mass that can become invasive infiltrating the chest wall and muscle, in some instances spreading to adjacent lymph nodes, in these cases having a far worse prognosis. The causation of BIA-ALCL remains to be established, but it has been proposed that chronic infection and/or implant toxins may be involved. What is clear is that complete capsulectomy is required for treatment of BIA-ALCL, which for early-stage disease leads to cure, whereas chemotherapy is needed for advanced-stage disease, whereby improved results have been reported with the use of brentuximab. A worldwide database for BIA-ALCL and implants should be supported by local governments.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6501645/

Record 964

pmcid: PMC6900746

pmid: 31356659

doi: 10.1093/jmp/jhz010

title: Is There a Problem With False Hope?

authors

- Musschenga B

journal: The Journal of medicine and philosophy

journalAbbr: J Med Philos

year: 2019

pubdate: 2019 Jul 29

volume: 44

issue: 4

pages: 423-441

abstract: This article offers a general discussion of the concept of false hope. Its ultimate aim is to clarify the meaning and the relevance of that concept for medicine and medical research. In the first part (Sections I-V), the concept of hope is discussed. I argue that hope is more than a combination of a desire and a belief about the probability that the desire will be fulfilled. Imagination and anticipation are as well components of hope. I also discuss if hope implies orientation to action. In the second part (Sections VI-VIII), I examine the concept of false hope. I show that hope is false if it cannot be justified epistemically. There is, I argue, an intimate relation

between false hope and ignorance. Hope is justified-"realistic"-when the hoping person knows and accepts experts' judgement about the probability of hope fulfillment. I then argue that what matters for evaluating a person's hope is not only whether it is realistic, but also whether it is reasonable in the light of the aim and goals that a person strives for in (the remainder of) his life. Part three (Sections IX and X) goes into the question of what is morally wrong about having or causing false hope. In the fourth part (Sections XI and XII), the relevance is shown of the insights from the previous parts for the debate on false hope in medicine and clinical research.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6900746/

Record 965

pmcid: PMC6343164

pmid: 30729125

doi: 10.1155/2019/5129263

title: Kisspeptin Protein in Seminal Plasma Is Positively Associated with Semen Quality: Results from the MARHCS Study in Chongqing, China.

authors

- Zou P
- Wang X
- Chen Q
- Yang H
- Zhou N
- Sun L
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- Liu J
- Ao L
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- Cao J

journal: BioMed research international

journalAbbr: Biomed Res Int

year: 2019

pubdate: 2019

volume: 2019

issue:

pages: 5129263

abstract: OBJECTIVES: To study the associations between kisspeptin levels in seminal plasma and blood plasma and semen quality. MATERIALS AND METHODS: We conducted a male reproductive health survey in June 2014. A total of 666 volunteers were recruited from colleges in Chongqing, China. All volunteers completed a questionnaire including information on domestic characteristics and some potential confounders. We tested the kisspeptin levels in both blood and seminal plasma. Total seminal kisspeptin was calculated as the concentration of kisspeptin in seminal plasma multiplied by semen volume. Semen samples were tested according to the 2010 World Health Organization's (WHO) guidelines. Spearman correlation and multivariate linear regression were used to explore the association between kisspeptin concentrations in seminal plasma and blood plasma and semen quality. Potential confounders that were adjusted for included age, abstinence time, body mass index (BMI), grade, and smoking. RESULTS: The median of kisspeptin levels in seminal plasma was 60,000 times higher than kisspeptin in blood plasma (28.0 × 10 6 pg/ml versus 448.9 pg/ml). Each interquartile range (IQR) of kisspeptin in seminal plasma was associated with a 4.6% (95% confidence interval [CI]: 1.6%-7.6%) increase in sperm concentration. Each IQR of total kisspeptin was associated with a 7.7%

(95% CI: 4.4%-11.0%) increase in total sperm number and a 7.8% (95% CI: 4.0%-11.7%) increase in total motile sperm count. Kisspeptin levels were further classified into quartiles and Q1 was set as the reference level. Subjects in the high total kisspeptin group had 57.5% (95% CI: 33.2%-86.2%) higher total sperm number than the reference group. CONCLUSION: The positive association between kisspeptin levels in seminal plasma and semen quality supported an important role for the KISS1/GPR54 system in male reproductive health. Kisspeptin may be a potential marker of male reproductive health and an alternative strategy for treating infertility.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6343164/

Record 966

pmcid: PMC6862634

pmid: 31694315

doi: 10.3390/ijerph16214301

title: Knowledge towards Cervical and Breast Cancers among Industrial Workers: Results from a Multisite Study in Northern Vietnam.

authors

- Tran BX
- Vo T
- Dang AK
- Nguyen QN
- Nguyen CT
- Hoang CL
- Do KN
- Latkin CA
- Ho CSH
- Ho RCM

journal: International journal of environmental research and public health

journalAbbr: Int J Environ Res Public Health

year: 2019

pubdate: 2019 Nov 5

volume: 16

issue: 21

pages:

abstract: Breast and cervical cancer cases are rising among service and industrial women workers in Vietnam. We conducted a cross-sectional study among 287 workers in three factories in Hanoi and Bac Ninh from July to September 2018 to describe the knowledge of these cancers among industrial workers in Northern Vietnam using a structured questionnaire. Factors associated with knowledge of breast and cervical cancer were identified using generalized linear models (GLM). In our study, approximately one-third of participants believed breast cancer was caused by the lack of breastfeeding, exposure to pollution, and chemicals. Less than 50% knew about sexually transmitted infections that can cause cervical cancer or were aware of a vaccine for cervical cancer. Having one sexual partner within the last year was positively associated with having a higher score of knowledge for both diseases. Receiving a medical checkup within the last 12 months and seeking health information via the internet were related to greater breast cancer knowledge. Targeted education campaigns are needed to ensure proper knowledge and improve awareness of breast cancer and cervical cancer among industrial workers.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6862634/

Record 967

pmcid: PMC6942760

pmid: 31929804

doi: 10.1155/2019/7584983

title: Labial Adhesions Causing Recurrent Urinary-Tract Infections in an Elderly Woman.

authors

- Takemaru M
- Aramaki-Hattori N
- Tsue C
- Kishi K

journal: Case reports in medicine

journalAbbr: Case Rep Med

year: 2019

pubdate: 2019

volume: 2019

issue:

pages: 7584983

abstract: A 91-year-old postmenopausal woman with a prior history of two labial-adhesion separations suffered from recurrent urinary-tract infections. We were able to successfully treat her labial adhesions using surgery.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6942760/

Record 968

pmcid: PMC6773459

pmid: 31393563

doi: 10.1210/jc.2018-01880

title: Lavender Products Associated With Premature Thelarche and Prepubertal Gynecomastia: Case Reports and Endocrine-Disrupting Chemical Activities.

authors

- Ramsey JT
- Li Y
- Arao Y
- Naidu A
- Coons LA
- Diaz A
- Korach KS

journal: The Journal of clinical endocrinology and metabolism

journalAbbr: J Clin Endocrinol Metab

year: 2019

pubdate: 2019 Nov 1

volume: 104

issue: 11

pages: 5393-5405

abstract: CONTEXT: Previous case reports associated prepubertal gynecomastia with lavender-containing fragrances, but there appear to be no reports of premature thelarche. OBJECTIVE: To add to a case series about lavender-fragranced product use and breast growth in children and to measure endocrine-disrupting chemical activity of essential oil components. DESIGN, SETTING, AND PATIENTS: Patients experiencing premature thelarche or prepubertal gynecomastia with continuous exposure to lavender-fragranced products were evaluated in the pediatric endocrinology departments of two institutions. Mechanistic in vitro experiments using eight components of lavender and other essential oils were performed at National Institute of Environmental Health Sciences. MAIN OUTCOME MEASURES: Case reports and in vitro estrogen and androgen receptor gene expression activities in human cell lines with essential oils. RESULTS: Three prepubertal girls and one boy with clinical evidence of estrogenic action and a history of continuous exposure to lavender-containing fragrances were studied. Breast growth dissipated in all patients with discontinuation of the fragranced products. Some of the components tested elicited estrogenic and antiandrogenic properties of varying degrees. CONCLUSION: We report cases of premature thelarche that resolved upon cessation of lavender-containing fragrance exposure commonly used in Hispanic communities. The precise developmental basis for such conditions could be multifactorial. In vitro demonstration of estrogenic and antiandrogenic properties of essential oil components suggests essential oils in these cases could be considered a possible source and supports a possible link with idiopathic prepubertal breast development. Whether the level of lavender oil estrogenic potency is sufficient to cause these effects is unknown.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6773459/

Record 969

pmcid: PMC6839758

pmid: 31772867

doi: 10.7759/cureus.5897

title: Liposuction in Gynecomastia: An Assessment of the Suction-assisted Arthroscopic Shaver Versus Open Disc Excision Techniques.

authors

- Akhtar A
- Eitezaz F
- Rashid M
- Khan I
- Malik SA

journal: Cureus

journalAbbr: Cureus

year: 2019

pubdate: 2019 Oct 12

volume: 11

issue: 10

pages: e5897

abstract: Introduction Gynecomastia is a common problem of the male breasts, which imposes a great psychological burden on patients. It is mostly bilateral and frequently asymmetrical. Surgical management of gynecomastia has undergone significant changes over the past few decades. Currently, the predominant mode of treatment includes liposuction of the fibro-fatty tissue either alone or in combination with the removal of the glandular tissue by the open excision technique or arthroscopic shaver. This study aims to compare both techniques in terms of hematoma formation, nipple necrosis, reoperation, contour irregularities, acceptability of scarring, asymmetry, and patient satisfaction. Methods The study has been conducted at Shifa International Hospital, Islamabad, from May 2018 to September 2019. Sixty patients were included in the study. All the

patients had bilateral gynecomastia and Simon's Grade II-A or II-B. The study sample was divided into two equal groups. Group A underwent liposuction combined with open disc excision while Group B underwent liposuction coupled with disc excision via suction-assisted arthroscopic shaver. Postoperatively, all the patients received follow-up for a minimum period of six months. Results In a cohort of 60 patients, the mean age was 25.76±5.38 years. There were minor differences noted in terms of hematoma formation, nipple necrosis, rates of re-operation, and contour irregularities between open disc excision and arthroscopic disc excision, respectively (p-value > 0.05). About eight patients reported asymmetry in open disc excision as compared to 10 in arthroscopic disc excision. The acceptability of scarring was reported as equal in both groups. Mean patient satisfaction was based on the visual analog scale (VAS) scale was 8.25 in both groups. No statistical difference regarding patient satisfaction was noted in both groups (p-value 0.126). Conclusion Our study concludes that arthroscopic shaver-assisted disc excision despite being a novel and minimally invasive technique does not hold superiority over conventional open disc excision for the management of gynecomastia. Furthermore, in a developing country like Pakistan, there is a lack of expertise with the procedure and a need for more training among plastic surgeons.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6839758/

Record 970

pmcid: PMC6545491

pmid: 31275800

doi: 10.3205/iprs000136

title: Long-term clinical results and quality of life in patients undergoing autologous fat transplantation for breast augmentation using the BEAULI™ protocol.

authors

- Kwiatkowska K
- Krapohl BD
- Tanzella U
- Ueberreiter K

journal: GMS Interdisciplinary plastic and reconstructive surgery DGPW

journalAbbr: GMS Interdiscip Plast Reconstr Surg DGPW

year: 2019

pubdate: 2019

volume: 8

issue:

pages: Doc10

abstract: Introduction: Autologous fat transplantation for breast augmentation has become increasingly interesting for patients and surgeons but only a few standardized procedures are available. BEAULI™ (Berlin Autologous Lipotransfer) protocol provides a suitable method with a standardized protocol. The aim of the study was to trace the 5-year long-term results after breast enlargement using the BEAULI™ protocol and the determination of changes in quality of life in relation to the intervention. Patients and methods: The study included non-smoking, currently non-pregnant women from the first BEAULI™ study (2007-2010), who were operated only for aesthetic reasons. BMI values, the jugulum nipple distance (JND), the breast base, and the maximum breast circumference were determined. The patients answered also a questionnaire with 30 questions on the postoperative quality of life. Results: The results measured after 6 months remained constant over 5 years. There was an average increase of the JND by 1.8 cm or 9.5%, and a widening of the base by 1.2 cm or 8.8%, and of breast circumference by 4.4 cm or 24%. The patients' quality of life, especially self-confidence and acceptance of their own body, has improved significantly after the operation. Discussion: The satisfying 5-year long-term results and low complication rate are two big advantages of the BEAULI™ method. The option to use autologous fat transplantation for another purpose like for reconstruction of breasts after a mastectomy

increases the attractiveness of this method. Conclusions: Fat transplantation for breast enlargement using BEAULI™ is a high-quality method with good results, and it is an alternative to silicone implants or other autologous tissue transplantations. Patients are satisfied with the BEAULI™ protocol, the complication rate is small, and natural results are achieved with moderate scars.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6545491/

Record 971

pmcid: PMC6370120

pmid: 30653136

doi: 10.1097/MD.0000000000014116

title: Madelung disease: A case report.

authors

- Gao H
- Xin ZY
- Yin X
- Zhang Y
- Jin QL
- Wen XY

journal: Medicine

journalAbbr: Medicine (Baltimore)

year: 2019

pubdate: 2019 Jan

volume: 98

issue: 3

pages: e14116

abstract: RATIONALE: Madelung disease (MD), a rarely reported disease, also known as benign symmetric lipomatosis, a disorder resulting from alcoholic abuse. It's largely under-recognized and under-reported, possibly because of unawareness of the condition by physicians. PATIENT CONCERNS: A 45-year-old Chinese man presented with intermittent fatigue and abdominal distension and progressive bilateral breast enlargement. He has been a heavy drinker for ten years before onset of the disease with an average daily alcohol intake of more than 120 g/day. DIAGNOSIS: Due to the patient's symptoms, laboratory test results, radiographic findings, he was diagnosed with MD. INTERVENTIONS: We treated him with abstinence from alcohol and supportive therapy. OUTCOMES: The patient is now in stable condition, with improvement in symptoms during follow-up. LESSONS: Doctors, confronted with progressive bilateral breast enlargement in a patient with alcoholic liver disease, should be aware of the underreported MD. Recognition of this syndrome could help doctors establish diagnosis and emphasize the importance of alcohol abstinence as the mainstay of management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6370120/

Record 972

pmcid: PMC6430715

pmid: 30903856

doi: 10.1016/j.ijscr.2019.02.028

title: Male DCIS diagnosed after use of over-the-counter hormonal supplement.

authors

- Agbroko SO
- Rojas KE
- Manasseh DM
- Borgen P

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2019

pubdate: 2019

volume: 57

issue:

pages: 60-62

abstract: INTRODUCTION: Male breast cancer is a rare disease. Although epidemiologic and genetic factors are associated with male breast cancer, hormonal factors may also play a role. CASE PRESENTATION: We report the case of a 39-year-old BRCA negative male patient taking a sexual performance enhancement supplement who presented with worsening asymmetric gynecomastia and unilateral spontaneous bloody nipple discharge and was found to have ductal carcinoma in-situ. DISCUSSION: The altered cellular environment related to the hormone contents of the supplement coincided with the rapid worsening of his gynecomastia and may have played a role in the development of the ductal carcinoma in-situ, or growth of an existing focus. CONCLUSION: The use of hormonal male enhancement supplements can lead to higher levels of androgens in users. It is possible for this altered hormonal environment to cause the growth of tumor or promote the progression of an existing focus.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6430715/

Record 973

pmcid: PMC6760881

pmid: 31572633

doi: 10.7759/cureus.5248

title: Male Gender Identity and Reversible Hypokalemic Hypertension in a 46,XX Child with 11-Beta-Hydroxylase Deficiency Congenital Adrenal Hyperplasia.

authors

- Goyal A
- Boro H
- Khadgawat R

journal: Cureus

journalAbbr: Cureus

year: 2019

pubdate: 2019 Jul 26

volume: 11

issue: 7

pages: e5248

abstract: Steroid 11-beta-hydroxylase deficiency is a relatively rare form of congenital adrenal hyperplasia (CAH). We describe the case of a 46,XX child, reared as a male, who first presented to us at the age of three years with features of peripheral precocity and hypokalemic hypertension. Based on the clinical and biochemical profile, a diagnosis of 11-beta-hydroxylase deficiency CAH was established, and physiological glucocorticoid

replacement was begun. Both hypertension and hypokalemia improved with glucocorticoid supplementation, and at eight years of age, antihypertensives were successfully withdrawn. Regression of left ventricular hypertrophy was also noted at this time. In keeping with the male gender identity, the child underwent hysterectomy, oopherectomy and breast reduction surgery at 13 years of age. We conclude that both hypertension and end-organ damage due to 11-beta-hydroxylase CAH may get reversed following optimal glucocorticoid treatment. Detailed genital examination at birth may help in early diagnosis of this rare disorder, thereby preventing the deleterious consequences of longstanding mineralocorticoid excess.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6760881/

Record 974

pmcid: PMC6635272

pmid: 31277192

doi: 10.1097/MD.0000000000016351

title: Management of biochemical recurrence after radical prostatectomy for prostate cancer: A case report.

authors

- Shen J
- Zang S
- Yu X
- Zhao F
- Jiang P
- Zhong B
- Zhou H
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journal: Medicine

journalAbbr: Medicine (Baltimore)

year: 2019

pubdate: 2019 Jul

volume: 98

issue: 27

pages: e16351

abstract: RATIONAL: How to manage patients with prostate cancer (PCa) with biochemical recurrence (BCR) following primary curative treatment is a controversial issue. Multiple disciplinary team (MDT) mechanism may propose an appropriate treatment plan for patients and can effectively improve patient prognosis and survival, reduce patient diagnosis and treatment waiting time, and greatly improve patient satisfaction. PATIENT CONCERNS: Here, we presented a case of a 77-year-old man with a persistently elevated serum level of prostate-specific antigen (PSA), who had a history of radical prostatectomy (RP) and of 9 years endocrine therapy. DIAGNOSES: Castration-resistant prostate cancer and locally recurrent prostate cancer. INTERVENTIONS: Androgen-deprivation therapy was first utilized 2 months after RP, due to the consideration of BCR on May 5, 2007. And during the next 9 years, he was treated with different endocrine agents but failed to maintain serum levels of PSA stable. Finally, the MDT suggested patient to perform salvage radiation therapy (SRT). Under MDT mechanism, we avoid secondary surgery, so as to reduce the patients' mental suffering and cost of patient care. OUTCOMES: EPIC26 scale assessment revealed leak-free urine, good urine control, no defecation abnormalities or blood in the stool, no breast tenderness and breast enlargement significantly improved. The patient now has no adjuvant therapy, including endocrine therapy. The patient achieved good prognosis through local RT. LESSONS: Pelvic SRT for patients with locally recurrent PCa may restore the same radical effect as RP. And more importantly, MDT mechanism plays an important role in making the most appropriate decisions for patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6635272/

Record 975

pmcid: PMC6432996

pmid: 30949280

doi:

title: Optimal Inframammary Fold Incision Length Based on Implant Volume for Breast Enlargement: A Cadaveric Study.

authors

- Muresan C
- Ford MM
- Anderson EW
- Lee TJ
- Hiller AR
- Kachare SD
- Wilhelmi BJ

journal: Eplasty

journalAbbr: Eplasty

year: 2019

pubdate: 2019

volume: 19

issue:

pages: e7

abstract: Purpose: Breast augmentation is the most commonly performed aesthetic operation in the Unites States annually. With the increasing popularity of gel implants, optimal incisional length for specific implant volumes becomes a factor to consider. Our study is the first, to date, to measure optimal incisional length for increasing Mentor smooth, round, moderate plus silicone implant volumes. Method: Three cadaver breasts were dissected in our anatomy laboratory. After dissection of a subpectoral pocket via an inframammary fold incision, time of implant insertion was measured for increasing volumes with the use of increasing incisional site lengths. Results: Values for increased incisions are as follows: 2.5-cm incision facilitated 100-cm 3 implant (average time [AvgT] 76 seconds); 3-cm incision 100-cm 3 implant (AvgT 32 seconds), 200 cm 3 (AvgT 84.33 seconds); 3.5-cm incision 100-cm 3 implant (AvgT 13.00 seconds), 200 cm 3 (AvgT 22.00 seconds), 300 cm 3 (AvgT 33.67 seconds); 4-cm incision 100-cm 3 implant (AvgT 5.67 seconds), 200 cm 3 (AvgT 11.33 seconds), 300 cm 3 (AvgT 21.33 seconds), 400 cm 3 (AvgT 26 seconds); 4.5-cm incision 100-cm 3 implant (AvgT 5 seconds), 200 cm 3 (AvgT 15.17 seconds), 300 cm 3 (AvgT 19.67 seconds), 400 cm 3 (AvgT 26 seconds), 500 cm 3 (AvgT 39.67 seconds), 600 cm 3 (AvgT 59.33 seconds), 700 cm 3 (AvgT 78.67 seconds); 5-cm incision 100-cm 3 implant (AvgT 1 second), 200 cm 3 (AvgT 3.67 seconds), 300 cm 3 (AvgT 8 seconds), 400 cm 3 (AvgT 13 seconds), 500 cm 3 (AvgT 19.33 seconds), 600 cm 3 (AvgT 23.33 seconds), 700 cm 3 (AvgT 28 seconds), 800 cm 3 (AvgT 33 seconds). Conclusion: Based on our results, the optimal incision size for increasing Mentor smooth, round, moderate plus volumes is as follows: 2.5 cm for 100-cm 3 implant, 3 cm for 200-cm 3 implant, 3.5 cm for 300-cm 3 implant, 4 cm for 400-cm 3 implant, 4.5 cm for 500- to 700-cm 3 implant, and 5 cm for 800-cm 3 implant. This guideline can be used to provide the most aesthetic results without tissue compromise.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6432996/

Record 976

pmcid: PMC6737443

pmid: 31538032

doi: 10.25259/JCIS_45_2019

title: Ovotesticular Disorder of Sex Development: An Unusual Presentation.

authors

- Özdemir M
- Kavak RP
- Yalcinkaya I
- Guresci K

journal: Journal of clinical imaging science

journalAbbr: J Clin Imaging Sci

year: 2019

pubdate: 2019

volume: 9

issue:

pages: 34

abstract: Disorder of sex development is an inclusive term that refers to any problem where the genital organ is atypical in relation to chromosomes or gonads. Ovotesticular disorder of sex development, which is formerly known as "true hermaphroditism," is the most rare form among all disorders of sex development in humans. It is characterized by the simultaneous presence of both ovarian and testicular tissues in the same individual and characteristically presents with ambiguous genitalia in neonates or infants. Herein, we present an unusual case of a 19-year-old individual with phenotypically nearly normal male genitalia who presented with the complaint of bilateral breast enlargement.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6737443/

Record 977

pmcid: PMC6443115

pmid: 30753468

doi: 10.1093/humrep/dez008

title: Parental time to pregnancy, medically assisted reproduction and pubertal development in boys and girls.

authors

- Ernst A
- Lauridsen LLB
- Brix N
- Arah OA
- Olsen J
- Olsen LH
- Ramlau-Hansen CH

journal: Human reproduction (Oxford, England)

journalAbbr: Hum Reprod

year: 2019

pubdate: 2019 Apr 1

volume: 34

issue: 4

pages: 724-732

abstract: STUDY QUESTION: Does parental fertility, measured by time to pregnancy (TTP), or use of medically assisted reproduction (MAR) affect pubertal development in the offspring? SUMMARY ANSWER: Neither TTP nor type of MAR treatment had clinically relevant implications for mean age at achieving individual pubertal milestones or overall timing of puberty in boys and girls. WHAT IS KNOWN ALREADY: Parental TTP and MAR have been associated with impaired semen quality in adult sons. Timing of puberty reflects earlier signals of reproductive health, but it remains unclear whether parental fertility or MAR affects pubertal development, especially in the growing generation of children conceived by IVF or ICSI. STUDY DESIGN, SIZE, DURATION: In this study, 15 819 children born by mothers in the Danish National Birth Cohort from 2000 to 2003 participated in a nationwide puberty cohort (participation rate = 70%). Parental TTP and use of MAR were reported by mothers in early pregnancy and children's pubertal development data was self-recorded in web-based questionnaires from 11 years of age and 6 monthly throughout puberty (2012-2018). PARTICIPANTS/MATERIALS, SETTING, METHODS: Pubertal development in children (of planned pregnancies, n = 13 285) born by untreated subfecund (TTP: 6-12 months) (n =2038), untreated severely subfeund (TTP: >12 months) (n = 1242), treated subfecund (n = 230) and treated severely subfecund (n = 1234) parents were compared to children born to more fertile parents (TTP: ≤5 months). We estimated mean monthly differences in mean age at achieving individual pubertal milestones (i.e. age at menarche, voice break, first ejaculation and Tanner stages 2, 3, 4 and 5 for breast or genital development and pubic hair growth) and a combined indicator of timing of puberty. Further, we compared mean age at achieving the individual pubertal milestones in children born by use of IVF or ICSI (n = 480) with children born by controlled ovarian stimulation or ovulation induction with or without intrauterine insemination (n = 902). MAIN RESULTS AND THE ROLE OF CHANCE: We found tendencies towards slightly later mean age at male pubertal timing and slightly earlier mean age at female pubertal timing among children born by untreated subfecund, treated subfecund, untreated severely subfecund and treated severely subfecund parents. There were no specific patterns with increasing TTP, use of MAR nor type of MAR treatment, and the magnitude of the mean differences for individual milestones and overall timing of puberty were small, i.e. 0.9 months (95% CI: -1.0; 2.8) for first ejaculation and -0.5 months (95% CI: -2.0; 1.0) months for age at menarche in boys and girls, respectively, born by treated severely subfecund parents when compared with children born by more fertile parents. LIMITATIONS, REASONS FOR CAUTION: Non-differential misclassification of the self-reported information on parental TTP and pubertal development in the offspring may serve as an alternative explanation of the findings, possibly biasing the estimates towards the null. The information on pubertal development was collected from around 11 years of age and onwards. WIDER IMPLICATIONS OF THE FINDINGS: This study adds to the growing body of literature suggesting only limited harmful effects of parental subfecundity and MAR on offspring's long-term growth and development. STUDY FUNDING/COMPETING INTEREST(S): This work was supported by the Danish Council for Independent Research [DFF 4183-00152]; and the Faculty of Health at Aarhus University. The authors have no financial relationships or competing interests to disclose.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6443115/

Record 978

pmcid: PMC6378315

pmid: 30804790

doi: 10.3389/fphar.2019.00104

title: Pelvic Ultrasound in Diagnosing and Evaluating the Efficacy of Gonadotropin-Releasing Hormone Agonist Therapy in Girls With Idiopathic Central Precocious Puberty.

authors

- Yu HK
- Liu X
- Chen JK
- Wang S
- Quan XY

journal: Frontiers in pharmacology

journalAbbr: Front Pharmacol

year: 2019

pubdate: 2019

volume: 10

issue:

pages: 104

abstract: Background and Objective: Idiopathic central precocious puberty (ICPP) is characterized by early pubertal changes, the acceleration of growth velocity, and rapid bone maturation that often results in reduced adult height. Gonadotrophin-releasing hormone agonist (GnRHa) is currently considered to be an effective therapeutic agent. At present, GnRH stimulation test is adopted as a gold standard for the diagnosis of ICPP and the efficacy evaluation of GnRHa therapy. However, it is difficult to operate in practice due to the cumbersome procedures and multiple blood samples required. This study was conducted to establish the value of pelvic ultrasound in diagnosing ICPP and evaluating the efficacy of GnRHa therapy. Materials and Methods: One hundred and twenty-two girls with ICPP (ICPP group) were enrolled in the study. Pelvic ultrasound and levels of luteinizing hormone (LH) and follicle-stimulating hormone (FSH) were examined before and after GnRHa therapy for 3 months. Eighty normal prepubertal girls were enrolled as the control group. The difference in pelvic ultrasound parameters between the ICPP group before GnRHa therapy and the control group was compared by independent-sample t -test, while paired t -test for ICPP group before and after GnRHa therapy. Receiver operating characteristic (ROC) curve was used to explore the optimal pelvic ultrasound parameters for diagnosing ICPP. Pearson correlation analysis was performed between the pelvic ultrasound parameters and serum sexual hormone level. Results: The pelvic ultrasound parameters (length of the uterine body, anteroposterior diameter of the uterine body, transverse diameter of the uterine body, volume of the uterine body, uterine body-cervix ratio, length of the ovary, transverse diameter of the ovary, anteroposterior diameter of the ovary, volume of the ovary, number of increased follicles and maximum diameter of the follicle) in the ICPP group before GnRHa therapy were significantly larger than those of the control group ($P < 0.05$). All the above pelvic ultrasound parameters in the ICPP group were significantly decreased after GnRHa therapy compared with those before treatment ($P < 0.05$). The volume of the uterine body had the largest area under the ROC curve in differentiating between patients with ICCP and the control group. Pelvic ultrasound parameters were significantly correlated with serum sexual hormone levels ($P < 0.05$). Conclusion: This study indicates pelvic ultrasound is a simple and reliable tool to diagnose ICPP and evaluates the efficacy of GnRHa therapy by dynamically observing the morphology of internal genitalia. The volume of uterine body was the best ultrasound parameter to distinguish patients with ICPP from normal girls.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6378315/

Record 979

pmcid: PMC7525710

pmid: 33060971

doi: 10.1007/s12055-018-0684-5

title: Perforation of the left ventricle during dissection of the intramuscular left anterior descending coronary artery-delayed presentation.

authors

- Machiraju V
- Trivedi DP

journal: Indian journal of thoracic and cardiovascular surgery

journalAbbr: Indian J Thorac Cardiovasc Surg

year: 2019

pubdate: 2019 Jan

volume: 35

issue: 1

pages: 59-61

abstract: Dissecting out a deep intramuscular left anterior descending coronary (LAD) artery during coronary artery revascularization requires surgical experience. Inadvertent entry into the right ventricle had been reported during such dissection previously. However, entry into the left ventricle (LV) has not. Here, we present a case where such an entry occurred and was successfully repaired.The patient presented ten years later with delayed rupture of the LV.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7525710/

Record 980

pmcid: PMC6614958

pmid: 31341683

doi: 10.1155/2019/2012546

title: Pituitary Hyperplasia in Severe Primary Hypothyroidism: A Case Report and Review of the Literature.

authors

- Shukla P
- Bulsara KR
- Luthra P

journal: Case reports in endocrinology

journalAbbr: Case Rep Endocrinol

year: 2019

pubdate: 2019

volume: 2019

issue:

pages: 2012546

abstract: Pituitary hyperplasia is commonly present but remains largely undiagnosed in primary hypothyroidism. It is easily reversible with thyroid replacement therapy. If imaging is performed prior to biochemical evaluation, then patients may undergo pituitary surgery. We present the case of a 34-year-old female with profound primary hypothyroidism and secondary pituitary hyperplasia that resolved after thyroid hormone supplementation. We will discuss the current literature regarding pituitary hyperplasia in primary hypothyroidism in adults.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6614958/

Record 981

pmcid: PMC6541761

pmid: 31146195

doi: 10.1016/j.ijscr.2019.04.042

title: Prednisolone-induced virginal mammary hypertrophy: Case report.

authors

- Jabaiti S
- Fayyad L
- Isleem U

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2019

pubdate: 2019

volume: 59

issue:

pages: 140-143

abstract: INTRODUCTION: Virginal mammary hypertrophy (VMH) is a rare benign disorder of the breast characterized by excessive enlargement of one or both breasts. It usually presents during adolescence. Drug-induced VMH has been scarcely reported in case reports. Review of the literature showed that prednisolone-induced gigantomastia was reported in a single study on a 47-year-old female. In this study, a case of VMH in a 17-year-old girl following prednisolone treatment will be described. Clinical presentation, clinical findings, diagnostic work-up, management and follow up are discussed. PRESENTATION OF CASE: A 17-year-old, single female presented to the plastic surgery department at our institute with progressive enlargement of both breasts, 2 months following treatment with prednisolone. Incisional biopsy excluded other breast differential diagnoses. The patient was managed surgically with bilateral mammaplasty with free nipple-areola graft. A total of 8.325 kg of breast tissue was resected. Follow-up 48 months postoperatively revealed good patient satisfaction with no recurrence of breast hypertrophy. DISCUSSION: Although the estrogen theory is regarded as the most credible explanation for VMH, several cases of drug-induced mammary hypertrophy have been reported. This study may be the first reported case of adolescent prednisolone-induced VMH. Mammaplasty is an accepted treatment despite its possible association with a higher recurrence rate. CONCLUSION: VMH may be a rare complication of prednisolone treatment. It should be considered in patients presenting with breast hypertrophy following steroid administration.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6541761/

Record 982

pmcid: PMC6680081

pmid: 31384229

doi: 10.1055/s-0039-1693438

title: Principles and Applications of Fat Grafting in Plastic Surgery.

authors

- Abu-Ghname A
- Perdanasari AT
- Reece EM

journal: Seminars in plastic surgery

journalAbbr: Semin Plast Surg

year: 2019

pubdate: 2019 Aug

volume: 33

issue: 3

pages: 147-154

abstract: Autologous fat transplantation has become increasingly popular in recent years. Its biocompatable properties and availability made it a widely used treatment modality for soft tissue augmentation and volume replacement in both reconstructive and aesthetic plastic surgery. Multiple protocols and clinical applications have been described in the literature, with wide variations in the harvesting, processing, and injection techniques. In this review, the authors will discuss the basic principles and clinical applications of fat grafting in plastic and reconstructive surgery. The article will then conclude with a discussion of fat grafting limitations as well as potential future applications, giving the reader a well-rounded understanding of autologous fat transfer.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6680081/

Record 983

pmcid: PMC6472847

pmid: 31019344

doi: 10.1590/0100-3984.2017.0135

title: Pseudoangiomatous stromal hyperplasia presenting as a tumor.

authors

- Nascimento TC
- Djahjah MC
- Carneiro AHPC
- de Oliveira AC
- Marchiori E

journal: Radiologia brasileira

journalAbbr: Radiol Bras

year: 2019

pubdate: 2019 Mar-Apr

volume: 52

issue: 2

pages: 128-129

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6472847/

Record 984

pmcid: PMC6355463

pmid: 30740498

doi: 10.1016/j.jdcr.2017.07.005

title: Pseudoangiomatous stromal hyperplasia presenting as an axillary mass in a postmenopausal woman undergoing in vitro fertilization.

authors

- Liu J
- Casady M
- Paral K
- Powers J

journal: JAAD case reports

journalAbbr: JAAD Case Rep

year: 2019

pubdate: 2019 Feb

volume: 5

issue: 2

pages: 159-161

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6355463/

Record 985

pmcid: PMC6962052

pmid: 31942085

doi: 10.1007/s10982-018-09341-3

title: Punishment as Moral Fortification and Non-Consensual Neurointerventions.

authors

- Theofilopoulou A

journal: Law and philosophy

journalAbbr: Law Philos

year: 2019

pubdate: 2019 Apr

volume: 38

issue: 2

pages: 149-167

abstract: The purpose of this paper is twofold. First, I defend and expand the Fortificationist Theory of Punishment (FTP). Second, I argue that this theory implies that non-consensual neurointerventions - interventions that act directly on one's brain - are permissible. According to the FTP, punishment is justified as a way of ensuring that citizens who infringe their duty to demonstrate the reliability of their moral powers will thereafter be able to comply with it. I claim that the FTP ought to be expanded to include citizens' interest in developing their moral powers. Thus, states must ensure that their citizens develop their moral reliability, not only because they must enforce their citizens' compliance with certain duties, but also because states have the duty to maintain the conditions for stability and satisfy their citizens' interest in developing their moral powers. According to this account of the FTP, if neurointerventions are the only or best way of ensuring that offenders can discharge their fortificational duties, states have strong reasons to provide these interventions.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6962052/

Record 986

pmcid: PMC6431563

pmid: 30473139

doi: 10.1016/j.kint.2018.08.034

title: Safety and cardiovascular efficacy of spironolactone in dialysis-dependent ESRD (SPin-D): a randomized, placebo-controlled, multiple dosage trial.

authors

- Charytan DM
- Himmelfarb J
- Ikizler TA
- Raj DS
- Hsu JY
- Landis JR
- Anderson AH
- Hung AM

- Mehrotra R
- Sharma S
- Weiner DE
- Williams M
- DiCarli M
- Skali H
- Kimmel PL
- Kliger AS
- Dember LM

journal: Kidney international

journalAbbr: Kidney Int

year: 2019

pubdate: 2019 Apr

volume: 95

issue: 4

pages: 973-982

abstract: The safety and efficacy of spironolactone is uncertain in end-stage renal disease. We randomized 129 maintenance hemodialysis patients to placebo (n=51) or spironolactone 12.5 mg (n=27), 25 mg (n=26), or 50 mg (n=25) daily for 36 weeks in a double-blind, placebo-controlled, multiple dosage trial to assess safety, tolerability and feasibility and to explore cardiovascular efficacy. The primary safety endpoints were hyperkalemia (potassium > 6.5 mEq/L) and hypotension requiring emergency department visit or hospitalization. Diastolic function was assessed by Doppler echocardiography. 125 participants (97%) completed dose escalation, with no significant difference in permanent study drug discontinuation between the groups (27.5% in placebo versus 16.7% in the combined spironolactone groups and 28% in the 50 mg group). Hyperkalemia frequency was similar between spironolactone and placebo (0.49 versus 0.50 events per patient-year) but demonstrated a significant linear trend due primarily to an increased event rate at the 50 mg dose (0.89 events per patient-year). The primary hypotension outcome was infrequent and similar with spironolactone and placebo (0.11 versus 0 events per patient-year). Gynecomastia was rare and did not differ significantly between groups. Change in diastolic function was similar with spironolactone and placebo. Spironolactone appears safe in carefully monitored maintenance hemodialysis patients, but did not affect cardiovascular parameters in this small study. Hyperkalemia occurs more frequently as dosage increases to 50 mg daily.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6431563/

Record 987

pmcid: PMC6939585

pmid: 31833813

doi: 10.1089/jwh.2018.7301

title: Sex and Gender Health Education Summit: Advancing Curricula Through a Multidisciplinary Lens.

authors

- McGregor AJ
- Chin EL
- Rojek MK
- Digre KB
- Lopez AM
- Jenkins K

- Johnston L
- Jenkins M

journal: Journal of women's health (2002)

journalAbbr: J Womens Health (Larchmt)

year: 2019

pubdate: 2019 Dec

volume: 28

issue: 12

pages: 1728-1736

abstract: Introduction: The Sex and Gender Health Education (SGHE) Summit was a national collaboration that engaged educational thought leaders from various health professions to advance curricula by integrating sex- and gender-based evidence into health education. Materials and Methods: The SGHE Summit was held over a 2.5-day period April 2018 at the University of Utah. Pre- and postsummit surveys assessed attitudinal and knowledge changes. Results: A total of 246 health care professionals and trainees from U.S. and International Institutions attended. One hundred fifty-seven presummit surveys and 115 postsummit surveys were completed. Postsummit beliefs: SGHE is critical to precision medicine (100%); it is essential to include female animals in preclinical research studies (96%); sex and gender concepts could be used to improve men's health (99%). A teaching tool summarizes the initial questions to consider in SGHE. Conclusion: The SGHE Summit was the first multiprofessional large-scale national effort focused on the integration of sex and gender knowledge into the education of all health professionals. Summit participants now represent a national network of educators and clinicians who recognize the centrality of sex and gender to health professionals' knowledge and practice. These educational efforts will ultimately ensure a more personalized health care delivery.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6939585/

Record 988

pmcid: PMC6862185

pmid: 31671702

doi: 10.3390/ijerph16214216

title: Sexual Functioning in Pregnant Women.

authors

- Fuchs A
- Czech I
- Sikora J
- Fuchs P
- Lorek M
- Skrzypulec-Plinta V
- Drosdzol-Cop A

journal: International journal of environmental research and public health

journalAbbr: Int J Environ Res Public Health

year: 2019

pubdate: 2019 Oct 30

volume: 16

issue: 21

pages:

abstract: Sexual activity during pregnancy is determined by emotional, psychosocial, hormonal, and anatomical factors and varies during trimesters. This work aimed to establish women's sexual activity during each trimester of pregnancy. A total of 624 women were included in the study and filled in the questionnaire three times, once during each trimester of pregnancy. The first part of the survey included questions about socio-demographic characteristics, obstetric history, and medical details of a given pregnancy. The second part was the Polish version of the female sexual function index (FSFI) questionnaire. Comparison of the mean scores for the overall sexual function of each trimester revealed clinically relevant sexual dysfunction in the second and third trimesters (mean values 25.9 ± 8.7 and 22.7 ± 8.7, respectively; $p < 0.01$). Women were most sexually active during their second trimester. In the first trimester of pregnancy, women were most likely to choose intercourse in the missionary position. Women with vocational education were characterized by the lowest and homogenous FSFI values. Total FSFI score depended on the martial status-the highest value pertained to married women (25.2 ± 6.9; $p = 0.02$).

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6862185/

Record 989

pmcid: PMC6552711

pmid:

doi: 10.1210/js.2019-SUN-207

title: SUN-207 Hot Flashes, Flushing, and Sweating in a Middle-Aged Man with Pituitary Macroadenoma.

authors

- Boddhula S
- Boddhula S
- Hughes J

journal: Journal of the Endocrine Society

journalAbbr: J Endocr Soc

year: 2019

pubdate: 2019 Apr 15

volume: 3

issue: Suppl 1

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6552711/

Record 990

pmcid: PMC6553423

pmid:

doi: 10.1210/js.2019-SUN-211

title: SUN-211 Male Pseudohermaphroditism Due to 5α-Reductase-2 Deficiency.

authors

- Selman-Geara A
- Selman-Geara A
- Cruz-Pineda O
- Selman-Fermin A
- Benitez-Camporro A

- Selman-Almonte A

journal: Journal of the Endocrine Society

journalAbbr: J Endocr Soc

year: 2019

pubdate: 2019 Apr 15

volume: 3

issue: Suppl 1

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6553423/

Record 991

pmcid: PMC6659039

pmid: 31360231

doi: 10.15420/ecr.2019.13.1

title: Testosterone and the Heart.

authors

- Kirby M
- Hackett G
- Ramachandran S

journal: European cardiology

journalAbbr: Eur Cardiol

year: 2019

pubdate: 2019 Jul

volume: 14

issue: 2

pages: 103-110

abstract: The development of a subnormal level of testosterone (T) is not universal in ageing men, with 75% of men retaining normal levels. However, a substantial number of men do develop T deficiency (TD), with many of them carrying a portfolio of cardiovascular (CV) risk factors, including type 2 diabetes (T2D) and the metabolic syndrome. TD increases the risk of CV disease (CVD) and the risk of developing T2D and the metabolic syndrome. The key symptoms suggesting low T are sexual in nature, including erectile dysfunction (ED), loss of night-time erections and reduced libido. Many men with heart disease, if asked, admit to ED being present; a problem that is often compounded by drugs used to treat CVD. A large number of studies and meta-analyses have provided evidence of the link between TD and an increase in CVD and total mortality. Patients with chronic heart failure (CHF) who have TD have a poor prognosis and this is associated with more frequent admissions and increased mortality compared with those who do not have TD. Conversely, in men with symptoms and documented TD, T therapy has been shown to have beneficial effects, namely improvement in exercise capacity in patients with CHF, improvement of myocardial ischaemia and coronary artery disease. Reductions in BMI and waist circumference, and improvements in glycaemic control and lipid profiles, are observed in T-deficient men receiving T therapy. These effects might be expected to translate into benefits and there are more than 100 studies showing CV benefit or improved CV risk factors with T therapy. There are flawed retrospective and prescribing data studies that have suggested increased mortality in treated men, which has led to regulatory warnings, and one placebo-controlled study demonstrating an increase in coronary artery non-calcified and total plaque volumes in men treated with T, which is open for debate. Men with ED and TD who fail to respond to

phosphodiesterase type 5 (PDE5) inhibitors can be salvaged by treating the TD. There are data to suggest that T and PDE5 inhibitors may act synergistically to reduce CV risk.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6659039/

Record 992

pmcid: PMC6797490

pmid: 31632203

doi: 10.1055/s-0039-1696985

title: The Evolution of Breast Implants.

authors

- Kaoutzanis C
- Winocour J
- Unger J
- Gabriel A
- Maxwell GP

journal: Seminars in plastic surgery

journalAbbr: Semin Plast Surg

year: 2019

pubdate: 2019 Nov

volume: 33

issue: 4

pages: 217-223

abstract: Breast augmentation remains one of the most commonly performed aesthetic procedures in the United States and worldwide. Throughout the last few decades, the implants used for this procedure have undergone significant advancements, which has allowed surgeons to provide safer and more aesthetically pleasing outcomes. This article discusses the history of breast implants since their invention in 1962. Particular emphasis is given to the evolution of silicone implants with its many challenges, which has resulted in the development of the currently used fourth- and fifth-generation devices. Knowledge of these advances will allow physicians to more critically evaluate their results, and also will encourage them to provide more up-to-date scientific data on these devices to further improve the clinical outcomes of their patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6797490/

Record 993

pmcid: PMC6406807

pmid: 30736442

doi: 10.3390/jcm8020209

title: The Optimal Indication for Testosterone Replacement Therapy in Late Onset Hypogonadism.

authors

- Shin YS
- Park JK

journal: Journal of clinical medicine

journalAbbr: J Clin Med

year: 2019

pubdate: 2019 Feb 7

volume: 8

issue: 2

pages:

abstract: : The use of testosterone replacement therapy (TRT) for late-onset hypogonadism (LOH) is increasing every year; however, the literature shows that many men are using testosterone (T) without a clear indication. Previous studies have estimated that up to 25% of men who receive TRT do not have their T tested prior to initiation of the therapy. Given the growing concern and need for proper TRT, clinicians need evidence-based information that informs them on the optimal indication for TRT in LOH patients. The diagnosis of LOH requires the presence of characteristic signs and symptoms, in combination with decreased serum total testosterone (TT). Based on the recent guidelines by the International Society for the Study of Aging Male (ISSAM), the European Association of Urology (EAU), the European Society of Endocrinology (ESE), the European Academy of Andrology (EAA), and the American Association of Urology (AUA), a TT of 250⁻350 ng/dL is the proper threshold value to define low T. The optimal indication for TRT in LOH is the presence of signs and symptoms of hypogonadism, and low T without contraindications for TRT.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6406807/

Record 994

pmcid: PMC6545764

pmid: 31236302

doi: 10.1155/2019/7946240

title: The Place of Reductive Surgery in the Management of Gestational Ulcerative Gigantomastia: A Case Report at Sourô Sanou Teaching Hospital.

authors

- Roland SO
- Cyprien Z
- Damien K
- Adama D
- Valentin K
- Nassirou Y
- Moussa B

journal: Case reports in surgery

journalAbbr: Case Rep Surg

year: 2019

pubdate: 2019

volume: 2019

issue:

pages: 7946240

abstract: INTRODUCTION: Gestational gigantomastia is a rare benign disease of unknown cause. We report a case of bilateral gigantomastia in pregnancy in order to locate the place of reductive surgery in its care which is quite controversial. CASE: A 25-year-old woman gravida 2 para 1 was referred for an exaggerated bilateral breast enlargement at 32-week gestation. The examination showed bilateral giant breasts with collateral venous circulation and trophic changes marked by the necrosis of the distal third of the mammary skin involving the nipple-areolar complex. She underwent a biopsy of the ulcerative breast tissue, and the histology report did not show a malignant cell. After active foetal lung maturation and induced delivery, a breast reductive surgery with

nipple plasty was performed 21 days postpartum. The postoperative course was marked by a period of lymphangitis. The cosmetic and psychological result was satisfactory at 6 months and at 18 months. CONCLUSION: Gestational gigantomastia is a benign disease that can simulate carcinomatous mastitis. The breast reductive plasty keeps its place in our context.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6545764/

Record 995

pmcid: PMC7032965

pmid: 32095480

doi: 10.12793/tcp.2019.27.4.127

title: The problem of medicating women like the men: conceptual discussion of menstrual cycle-dependent psychopharmacology.

authors

- Yum SK
- Yum SY
- Kim T

journal: Translational and clinical pharmacology

journalAbbr: Transl Clin Pharmacol

year: 2019

pubdate: 2019 Dec

volume: 27

issue: 4

pages: 127-133

abstract: While hormonal changes during the ovulatory cycles affect multiple body systems, medical management, including medication dosing remains largely uniform between the sexes. Little is known about sex-specific pharmacology in women. Although hormonal fluctuations of the normal menstruating process alters women's physiology and brain biochemistry, medication dosing does not consider such cyclical changes. Using schizophrenia as an example, this paper illustrates how a woman's clinical symptoms can change throughout the ovulatory cycle, leading to fluctuations in medication responses. Effects of sex steroids on the brain, clinical pharmacology are discussed. Effective medication dose may be different at different phases of the menstrual cycle. Further research is needed to better understand optimal treatment strategies in reproductive women; we present a potential clinical trial design for examining optimal medication dosing strategies for conditions that have menstruation related clinical fluctuations.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7032965/

Record 996

pmcid: PMC6775549

pmid: 30569789

doi: 10.1177/1557988318818280

title: The Pulsatile Gonadorelin Pump Induces Earlier Spermatogenesis Than Cyclical Gonadotropin Therapy in Congenital Hypogonadotropic Hypogonadism Men.

authors

- Zhang L
- Cai K
- Wang Y

- Ji W
- Cheng Z
- Chen G
- Liao Z

journal: American journal of men's health

journalAbbr: Am J Mens Health

year: 2019

pubdate: 2019 Jan-Feb

volume: 13

issue: 1

pages: 1557988318818280

abstract: The objective of this study was to compare the effect of pulsatile gonadorelin pump (PGP) and cyclical gonadotropin (human chorionic gonadotropin [HCG]/human menopausal gonadotropin [HMG]) therapy (CGT) on spermatogenesis in congenital hypogonadotropic hypogonadism (CHH) men. Twenty-eight azoospermic CHH males were included in this nonrandomized study. Ten received PGP and 18 received CGT. The primary endpoint was the earliest time spermatogenesis occurred during 24 months of treatment. Spermatogenesis time was significant earlier in the PGP group than the CGT group (median of 6 and 14 months, respectively, χ 2 = 6.711, p = .01). Spermatogenesis occurred in 90% of the PGP group and 83.3% of the CGT group and showed statistically insignificant difference in the superiority analysis and the no-inferior test. Contributing factors significant for spermatogenesis were previous HCG/or testosterone treatment and the peak serum luteinizing hormone level of triptorelin stimulation test at baseline. Although testis volume and penile length increased significantly from baseline, the differences between the two therapies were not significant. There was a tendency for high serum testosterone level, associated with more facial acne and breast tenderness in the CGT group. Skin allergic erythema scleroma was a common side effect of the PGP. In summary, PGP resulted in earlier spermatogenesis and more desirable testosterone levels than CGT.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6775549/

Record 997

pmcid: PMC6639989

pmid: 31367069

doi: 10.4103/iju.IJU_56_19

title: The resurgence of estrogens in the treatment of castration-resistant prostate cancer.

authors

- Moorthy HK
- Laxman Prabhu GG
- Venugopal P

journal: Indian journal of urology : IJU : journal of the Urological Society of India

journalAbbr: Indian J Urol

year: 2019

pubdate: 2019 Jul-Sep

volume: 35

issue: 3

pages: 189-196

abstract: Use of exogenous estrogens in manipulating the androgenestrogen equilibrium was one of the earliest

therapeutic strategies developed to treat prostate cancer which followed close on heels the discovery of hormone dependence of this tumor. Despite its well-documented benefit, estrogen therapy fell out of favor with the advent of other forms of androgen deprivation therapy (ADT) as the former registered a higher incidence of cardiovascular complications and poorer overall survival. Clearer understanding of the mechanism of action of estrogen coupled with the adoption of alternative routes of administration has triggered a renewed interest in estrogen therapy. Since then, many studies have not only proved the therapeutic benefit of estrogens but also explored the ways and means of minimizing the dreaded side effects deterring its use. Further, the fact that estrogen therapy offered a clear advantage of reduced cost of treatment over other treatments has led many countries to readopt it in the treatment of advanced prostatic cancer. We reviewed the published data on the use of estrogens in CRPC, which may affect its revival as an efficacious treatment option having minimal side effects, with modified dosage and route of administration. Estrogen therapy would be a less expensive option having equivalent or even better therapeutic effect than ADT in advanced carcinoma of prostate.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6639989/

Record 998

pmcid: PMC6536879

pmid: 31113185

doi: 10.5999/aps.2018.00472

title: The round-the-clock technique for correction of gynecomastia.

authors

- Tarallo M
- Di Taranto G
- Fallico N
- Ribuffo D

journal: Archives of plastic surgery

journalAbbr: Arch Plast Surg

year: 2019

pubdate: 2019 May

volume: 46

issue: 3

pages: 221-227

abstract: BACKGROUND: Gynecomastia is a common condition that can cause severe emotional and physical distress in both young and older men. Patients in whom symptomatic recalcitrant gynecomastia persists for a long time are potential candidates for surgery. METHODS: From January 2014 to January 2016, 15 patients underwent correction of gynecomastia through a single 3-mm incision at our institution. Only patients with true gynecomastia underwent surgery with this new technique. Through the small incision, sharp dissection was performed in a clockwise and counterclockwise direction describing two half-circles. Health-related quality of life and aesthetic outcomes were evaluated using a modified version of the Breast Evaluation Questionnaire (BEQ). RESULTS: The patients' average age was 23.5 years (range, 18-28 years), and their average body mass index was 23.2 kg/m2 (range, 19.2-25.3 kg/m2). One case was unilateral and 14 cases were bilateral. The weight of glandular tissue resected from each breast ranged from 80 to 170 g. No excess skin was excised. Bleeding was minimal. The mean operating time was 25 minutes (range, 21-40 minutes). No complications were recorded. All lesions were histologically benign. The patients' average score was 3.5 (on a 5-point Likert scale) in all domains of the BEQ for themselves and their partners. CONCLUSIONS: In this study, we demonstrated the safety and reliability of a new technique that allows mastectomy through an imperceptible 3-mm incision. We obtained high patient satisfaction scores using our surgical technique, and patients reported considerable improvement in their social, physical, and psychological well-being after surgery.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6536879/

Record 999

pmcid: PMC6559975

pmid: 31217704

doi: 10.14744/TurkPediatriArs.2019.04810

title: Transient endocrinologic problems in the newborn period.

authors

- Kurtoğlu S
- Direk G
- Tatlı ZU
- Hatipoğlu N

journal: Turk pediatri arsivi

journalAbbr: Turk Pediatri Ars

year: 2019

pubdate: 2019

volume: 54

issue: 1

pages: 3-12

abstract: Many transient endocrinologic disorders are frequently seen in newborn period. Early diagnosis and treatment is important for babies. In this article, transient endocrinopathy of newborn and relevant literature were reviewed. Blood sugar problems, especially adrenal insufficiency due to adrenal problems, thyroid problems such as transient hypotirotropinemia, are frequently encountered by physicians. Genital and urinary problems should be evaluated differently according to gender. Problems related to calcium metabolism, problems associated with water metabolism and endocrine skin problems are other problems. It is essential to know the normals of the hormones in the neonatal period in order to recognize them properly, to evaluate them properly and to interpret the tests correctly.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6559975/

Record 1000

pmcid: PMC6567756

pmid: 30958070

doi: 10.1177/0300060519840896

title: Treatment of gynecomastia with prednisone: case report and literature review.

authors

- Jin Y
- Fan M

journal: The Journal of international medical research

journalAbbr: J Int Med Res

year: 2019

pubdate: 2019 May

volume: 47

issue: 5

pages: 2288-2295

abstract: This study aimed to report a unique case of primary adrenal insufficiency that was accompanied by painful gynecomastia, which was resolved by treatment with prednisone. Enlargement of the left breast with continuous weakness and generalized nausea in a male was discovered 3 months before admission. Magnetic resonance imaging of the brain was normal 1 month before presentation. A physical examination revealed that the diameter of the left breast was 5 cm and the height was 3 cm. Laboratory investigations revealed hyponatremia, with a low serum cortisol level and an elevated prolactin level. Hyperprolactinemia was suspected because of adrenal deficiency that was directly or indirectly associated with increased prolactin levels. Thus, a diagnosis of hyperprolactinemia was confirmed. Ultrasonography of the left breast showed glandular tissue hyperplasia. In the present study, treating adrenal insufficiency with prednisone relieved both gynecomastia and hyponatremia. However, gynecomastia regression and hyponatremia resolution were observed when prednisone was stopped. Gynecomastia completely resolved by re-administering prednisone. Therefore, treating the underlying disease is essential so that prednisone can be given in a timely manner.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6567756/

Record 1001

pmcid: PMC6698440

pmid: 31440564

doi: 10.1016/j.jdcr.2019.06.006

title: Unilateral nipple hyperkeratosis.

authors

- Shah N
- Delost GR
- Sharma TR
- Scott JF

journal: JAAD case reports

journalAbbr: JAAD Case Rep

year: 2019

pubdate: 2019 Aug

volume: 5

issue: 8

pages: 709-712

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6698440/

Record 1002

pmcid: PMC6797485

pmid: 31632210

doi: 10.1055/s-0039-1697028

title: Update in Direct-to-Implant Breast Reconstruction.

authors

- Perdanasari AT
- Abu-Ghname A
- Raj S
- Winocour SJ

- Largo RD

journal: Seminars in plastic surgery

journalAbbr: Semin Plast Surg

year: 2019

pubdate: 2019 Nov

volume: 33

issue: 4

pages: 264-269

abstract: Implant-based reconstruction (IBR) remains the most commonly utilized breast reconstruction option for post-mastectomy patients. IBR can be approached as either a one-stage reconstruction or a two-stage reconstruction. Facilitated by improvements in surgical technology and advanced techniques, one-stage reconstruction, also known as direct-to-implant (DTI) reconstruction, involves the insertion of an implant at the time of mastectomy. The decision to pursue either a DTI or a two-stage reconstruction is based on several factors, including the patient's overall health, expected risk of postoperative complications, and associated costs to both the patient and hospital.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6797485/

Record 1003

pmcid: PMC6501472

pmid: 31106249

doi: 10.1177/2333794X19847923

title: Utilizing a Risk Factor Approach to Identify Potential Breastfeeding Problems.

authors

- Flagg J
- Busch DW

journal: Global pediatric health

journalAbbr: Glob Pediatr Health

year: 2019

pubdate: 2019

volume: 6

issue:

pages: 2333794X19847923

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6501472/

Record 1004

pmcid: PMC6023667

pmid: 29923976

doi: 10.1097/MD.0000000000010916

title: A rare case report of ovarian juvenile granulosa cell tumor with massive ascites as the first sign, and review of literature: Case report and review of literature.

authors

- Ma L
- Zhang L
- Zhuang Y
- Ding Y
- Chen J

journal: Medicine

journalAbbr: Medicine (Baltimore)

year: 2018

pubdate: 2018 Jun

volume: 97

issue: 25

pages: e10916

abstract: RATIONALE: Massive ascites as the first sign of ovarian juvenile granulosa cell tumor (JGCT) in an adolescent is an extremely rare, and its clinical features and treatment methods have not been well described. PATIENT CONCERNS: The clinical characteristics, diagnosis, and treatment methods in a 19-year-old girl who presented with massive abdominal distention and ascites was retrospectively reviewed. Abdominopelvic ultrasonography showed a large amount of ascites. The nature of ascites was exudate. All tumor markers were normal, but ascites and serum tumor CA125 levels were significantly increased. Abdominal CT showed left attachment area teratoma and right attachment area capsule solid change. DIAGNOSES: Histological and immunohistochemical results were compatible with JGCT. Based on the FIGO classification, the patient with only malignant ascites was categorized into stage IC. INTERVENTIONS: The patient underwent mass resection with salpingoophorectomy. Following the operation, she received 6 courses of adjuvant chemotherapy with Nedaplatin and Paclitaxel liposome. OUTCOMES: The patient was followed up postoperatively for 6 months to date without recurrence. LESSONS: We should be highly vigilant the JGCT with massive ascites as the first clinical manifestation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6023667/

Record 1005

pmcid: PMC6212147

pmid:

doi: 10.1097/01.GOX.0000547101.14714.b0

title: Abstract: Adolescent Breast Reduction and Gynecomastia Surgery: BMI Comparisons and Correlations.

authors

- Kaye AE
- Cattaneo CJ
- Tracy MM

journal: Plastic and Reconstructive Surgery Global Open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2018

pubdate: 2018 Sep

volume: 6

issue: 9 Suppl

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6212147/

Record 1006

pmcid: PMC6101308

pmid: 30121569

doi: 10.1136/bcr-2018-225735

title: Acute myeloid leukaemia presenting with bilateral breast masses.

authors

- Cheng KCA
- Li YL
- Lam T

journal: BMJ case reports

journalAbbr: BMJ Case Rep

year: 2018

pubdate: 2018 Aug 17

volume: 2018

issue:

pages:

abstract: A 46-year-old woman with a known history of acute myeloid leukaemia presented with bilateral breast masses with pain and itchiness. The breast masses were hard on palpation. Mammogram was unremarkable. Ultrasound showed multiple conglomerated masses of heterogeneous hyperechogenicity and hypoechogenicity throughout all quadrants of bilateral breasts. Pathology showed mononuclear cells, suggestive of breast leukaemic infiltration. She was treated with decitabine and platelet transfusion.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6101308/

Record 1007

pmcid: PMC6043751

pmid: 30050798

doi: 10.21037/tau.2018.01.06

title: Adjuvant androgen deprivation therapy for prostate cancer treated with radiation therapy.

authors

- Siddiqui ZA
- Krauss DJ

journal: Translational andrology and urology

journalAbbr: Transl Androl Urol

year: 2018

pubdate: 2018 Jun

volume: 7

issue: 3

pages: 378-389

abstract: Radiation therapy is a commonly used curative modality for prostate cancer. The addition of androgen deprivation therapy (ADT) increases the curative potential of prostate radiotherapy (RT) in multiple subsets of

patients. In addition to having an independent cytotoxic effect, current evidence suggests that androgen deprivation synergistically works with radiation therapy by preventing DNA repair. Given the wide-ranging toxicities of this therapy, clinicians must judiciously choose which patients may benefit from ADT and also consider the appropriate length of treatment. With recent advances in RT delivery, higher doses of radiation are currently used when compared with the dose used in historic trials, leading to the unanswered question of how RT dose interacts with ADT. Current and future clinical studies are attempting to further define the appropriate indications and patient populations for which ADT represents a clinically appropriate addition to prostate RT.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6043751/

Record 1008

pmcid: PMC6156700

pmid: 30263085

doi: 10.1016/j.radcr.2018.09.001

title: Bilateral diffuse tumorous pseudoangiomatous stromal hyperplasia treated with bilateral mastectomy in a 40-year-old woman.

authors

- Ko SY
- Koh MJ

journal: Radiology case reports

journalAbbr: Radiol Case Rep

year: 2018

pubdate: 2018 Dec

volume: 13

issue: 6

pages: 1271-1275

abstract: Pseudoangiomatous stromal hyperplasia (PASH) is a rare benign breast disease characterized by breast stromal proliferation mimicking low grade angiosarcoma. PASH is frequently detected as an incidental component coexisting with other breast lesions. However, it can also present as a single localized mass and is typically unilateral. Mammographic and sonographic findings are nonspecific and indistinguishable from those of benign lesions. We report an unusual case of PASH presenting with diffuse enlargement of the bilateral breasts in a 40-year-old woman treated with bilateral mastectomy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6156700/

Record 1009

pmcid: PMC5948190

pmid: 29752634

doi: 10.1007/s40800-018-0085-0

title: Bilateral Mastectomy as Radical Treatment of Gynecomastia Secondary to Antiretroviral Therapy in a Low-Income Setting: A Case Report.

authors

- Antunes M
- Schiavone M
- Pizzol D
- Di Gennaro F

- Ludovico R
- De Palma A

journal: Drug safety - case reports

journalAbbr: Drug Saf Case Rep

year: 2018

pubdate: 2018 May 11

volume: 5

issue: 1

pages: 21

abstract: Gynecomastia is a common finding in males, with an incidence that varies widely globally. In 10-25% of cases, it is caused by drugs. Its pathophysiologic mechanism includes exposure to exogenous estrogens and medications that cause hypogonadism, antiandrogenic effects and hyperprolactinemia. Gynecomastia is associated with exposure to antiretroviral therapy (ART), particularly efavirenz. Sometimes surgery may be required as treatment. We report a case of a 46-year-old man receiving ART presenting with a marked bilateral breast enlargement who underwent bilateral mastectomy as the only successful treatment in a low-income setting.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5948190/

Record 1010

pmcid: PMC6020282

pmid: 29971157

doi: 10.1192/bjo.2018.1

title: Bipolar disorder, comorbid anxiety disorders, gynecomastia and dental pain: case analysis with literature review.

authors

- Kaufman KR
- Babalola R
- Campeas M
- Coluccio M

journal: BJPsych open

journalAbbr: BJPsych Open

year: 2018

pubdate: 2018 May

volume: 4

issue: 3

pages: 137-141

abstract: UNLABELLED: Bipolar disorder with comorbid anxiety disorders frequently requires rational polypharmacy, including use of serotonergic psychotropics. These may result in adverse effects, influencing adherence, complicating treatment and confounding diagnoses. Serotonergic non-adherence is associated with discontinuation syndromes. In this complex case with an on/off/on/off design, both dose-dependent buspirone-induced gynecomastia and buspirone discontinuation syndrome with dental pain are reported. Clinicians and patients should consider these findings to maximise treatment adherence, minimise any unnecessary interventions and address unusual adverse effects. Since patients may not voluntarily disclose specific adverse effects and often do not acknowledge non-adherence, clinician-directed questions are required. This case

further emphasises the importance of medication and symptom timelines to guide determination of causation for adverse effects. Although findings from this case cannot be generalised, they suggest the need for continued clinician and patient education, as well as the benefit from detailed case reports. DECLARATION OF INTEREST: None.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6020282/

Record 1011

pmcid: PMC6147049

pmid: 30258937

doi: 10.1016/j.gendis.2018.05.001

title: Breast cancer development and progression: Risk factors, cancer stem cells, signaling pathways, genomics, and molecular pathogenesis.

authors

- Feng Y
- Spezia M
- Huang S
- Yuan C
- Zeng Z
- Zhang L
- Ji X
- Liu W
- Huang B
- Luo W
- Liu B
- Lei Y
- Du S
- Vuppalapati A
- Luu HH
- Haydon RC
- He TC
- Ren G

journal: Genes & diseases

journalAbbr: Genes Dis

year: 2018

pubdate: 2018 Jun

volume: 5

issue: 2

pages: 77-106

abstract: As the most commonly occurring cancer in women worldwide, breast cancer poses a formidable public health challenge on a global scale. Breast cancer consists of a group of biologically and molecularly heterogeneous diseases originated from the breast. While the risk factors associated with this cancer varies with respect to other cancers, genetic predisposition, most notably mutations in BRCA1 or BRCA2 gene, is an important causative factor for this malignancy. Breast cancers can begin in different areas of the breast, such as the ducts, the lobules, or the tissue in between. Within the large group of diverse breast carcinomas, there are

various denoted types of breast cancer based on their invasiveness relative to the primary tumor sites. It is important to distinguish between the various subtypes because they have different prognoses and treatment implications. As there are remarkable parallels between normal development and breast cancer progression at the molecular level, it has been postulated that breast cancer may be derived from mammary cancer stem cells. Normal breast development and mammary stem cells are regulated by several signaling pathways, such as estrogen receptors (ERs), HER2, and Wnt/β-catenin signaling pathways, which control stem cell proliferation, cell death, cell differentiation, and cell motility. Furthermore, emerging evidence indicates that epigenetic regulations and noncoding RNAs may play important roles in breast cancer development and may contribute to the heterogeneity and metastatic aspects of breast cancer, especially for triple-negative breast cancer. This review provides a comprehensive survey of the molecular, cellular and genetic aspects of breast cancer.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6147049/

Record 1012

pmcid: PMC6206369

pmid: 30187266

doi: 10.1007/s13244-018-0652-z

title: Breast implant-associated anaplastic large cell lymphoma: a pictorial review.

authors

- Chacko A
- Lloyd T

journal: Insights into imaging

journalAbbr: Insights Imaging

year: 2018

pubdate: 2018 Oct

volume: 9

issue: 5

pages: 683-686

abstract: Breast implant-associated anaplastic large cell lymphoma (BIA-ALCL) is a newly described and rare T-cell lymphoma of the breast. Since 2007, there have been 56 cases of confirmed BIA-ALCL in Australia and New Zealand. The incidence is believed to be on the rise as the prevalence of elective breast implantation increases. In 2016, the World Health Organization (WHO) classified BIA-ALCL as a recognised entity and emphasised the importance of surgical management of the disease. BIA-ALCL typically presents as a delayed, non-infective fluid collection around a textured breast implant or residual fibrous scar capsule. The mean age of presentation is 47 years, with an average time frame of 7.5 years following breast implantation. Although rare, BIA-ALCL is increasing in incidence. To avoid delays in diagnosis, radiologists should consider this form of lymphoma in the differential of any non-acute peri- or post-prosthetic effusion, and suggest cytological evaluation, so as not to miss this rare but important diagnosis. TEACHING POINTS: • BIA-ALCL is a newly described and rare T-cell lymphoma of the breast. • Since 2007, there have been 56 cases of confirmed BIA-ALCL in Australia and New Zealand. • BIA-ALCL presents as a delayed, non-infective fluid collection. • The effusion typically accumulates around a textured breast implant or residual fibrous capsule.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6206369/

Record 1013

pmcid: PMC6092150

pmid: 30123878

doi: 10.5152/ejbh.2018.3871

title: Breast Infection: A Review of Diagnosis and Management Practices.

authors

- Boakes E
- Woods A
- Johnson N
- Kadoglou N

journal: European journal of breast health

journalAbbr: Eur J Breast Health

year: 2018

pubdate: 2018 Jul

volume: 14

issue: 3

pages: 136-143

abstract: Mastitis is a common condition that predominates during the puerperium. Breast abscesses are less common, however when they do develop, delays in specialist referral may occur due to lack of clear protocols. In secondary care abscesses can be diagnosed by ultrasound scan and in the past the management has been dependent on the receiving surgeon. Management options include aspiration under local anesthetic or more invasive incision and drainage (I&D). Over recent years the availability of bedside/clinic based ultrasound scan has made diagnosis easier and minimally invasive procedures have become the cornerstone of breast abscess management. We review the diagnosis and management of breast infection in the primary and secondary care setting, highlighting the importance of early referral for severe infection/breast abscesses. As a clear guideline on the management of breast infection is lacking, this review provides useful guidance for those who rarely see breast infection to help avoid long-term morbidity.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6092150/

Record 1014

pmcid: PMC6039723

pmid: 30008556

doi:

title: Can Judges Ration with Compassion? A Priority-Setting Rights Matrix.

authors

- Newdick C

journal: Health and human rights

journalAbbr: Health Hum Rights

year: 2018

pubdate: 2018 Jun

volume: 20

issue: 1

pages: 107-120

abstract: How should courts supervise health service resource allocation? Although practice varies widely, four broad approaches can be represented on a matrix comparing, on two axes, (a) individual-community rights and (b) substantive-procedural remedies. Examples from each compartment of the matrix are discussed and, although the community-procedural approach is recommended as a general rule, a range of other responses within the matrix may also be desirable.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6039723/

Record 1015

pmcid: PMC5865915

pmid: 29616171

doi: 10.1097/GOX.0000000000001673

title: Combining Breast Reduction Techniques to Treat Gigantomastia in Ghana.

authors

- Scheefer MF
- Agbenorku P
- Hoyte-Williams PE
- Farhat B
- Goodwin IA
- Rockwell WB

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2018

pubdate: 2018 Feb

volume: 6

issue: 2

pages: e1673

abstract: In this presentation of 2 consecutive cases of symptomatic juvenile breast hypertrophy in Ghana, we review the patient presentation, workup, and discuss outcomes following a combined technique of inferior pedicle stump with free nipple graft reduction mammoplasty. Surgical goals for treatment of gigantomastia were 2-fold: to resect adequate tissue to obtain symptomatic relief with improved quality of life, while avoiding a flat, boxy-appearing breast shape.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5865915/

Record 1016

pmcid: PMC5904990

pmid: 29665850

doi: 10.1186/s13058-018-0954-6

title: Common genetic variation and novel loci associated with volumetric mammographic density.

authors

- Brand JS
- Humphreys K
- Li J
- Karlsson R
- Hall P
- Czene K

journal: Breast cancer research : BCR

journalAbbr: Breast Cancer Res

year: 2018

pubdate: 2018 Apr 17

volume: 20

issue: 1

pages: 30

abstract: BACKGROUND: Mammographic density (MD) is a strong and heritable intermediate phenotype of breast cancer, but much of its genetic variation remains unexplained. METHODS: We conducted a genetic association study of volumetric MD in a Swedish mammography screening cohort (n = 9498) to identify novel MD loci. Associations with volumetric MD phenotypes (percent dense volume, absolute dense volume, and absolute nondense volume) were estimated using linear regression adjusting for age, body mass index, menopausal status, and six principal components. We also estimated the proportion of MD variance explained by additive contributions from single-nucleotide polymorphisms (SNP-based heritability [h 2 SNP]) in 4948 participants of the cohort. RESULTS: In total, three novel MD loci were identified (at $P < 5 \times 10 - 8$): one for percent dense volume (HABP2) and two for the absolute dense volume (INHBB, LINC01483). INHBB is an established locus for ER-negative breast cancer, and HABP2 and LINC01483 represent putative new breast cancer susceptibility loci, because both loci were associated with breast cancer in available meta-analysis data including 122,977 breast cancer cases and 105,974 control subjects ($P < 0.05$). h 2 SNP (SE) estimates for percent dense, absolute dense, and nondense volume were 0.29 (0.07), 0.31 (0.07), and 0.25 (0.07), respectively. Corresponding ratios of h 2 SNP to previously observed narrow-sense h 2 estimates in the same cohort were 0.46, 0.72, and 0.41, respectively. CONCLUSIONS: These findings provide new insights into the genetic basis of MD and biological mechanisms linking MD to breast cancer risk. Apart from identifying three novel loci, we demonstrate that at least 25% of the MD variance is explained by common genetic variation with h 2 SNP /h 2 ratios varying between dense and nondense MD components.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5904990/

Record 1017

pmcid: PMC6193680

pmid: 30335816

doi: 10.1371/journal.pone.0205737

title: Complexities and dilemmas in community consultation on the design of a research project logo in Malawi.

authors

- Nyirenda D
- Gooding K
- Lora W
- Kumwenda M
- McMorrow M
- Everett D
- Desmond N

journal: PloS one

journalAbbr: PLoS One

year: 2018

pubdate: 2018

volume: 13

issue: 10

pages: e0205737

abstract: BACKGROUND: Community engagement on research design is widely highlighted as an important

approach for ethical research. This article reports the experience of consulting with communities on the logo used for an influenza study in Malawi. The logo was designed for use on badges worn by study researchers, participant information sheets and other project documents, and could affect perceptions of the study and consequent engagement in the research. METHODS: Four focus group discussions were conducted with populations targeted by the influenza study: pregnant women, people with HIV, mothers and community members. The focus groups incorporated a participatory matrix exercise focusing on key themes emerging from the discussions such as: attractiveness, comprehension, acceptability and suggestions for improvement. Findings from the focus groups were analyzed according to these key themes. RESULTS: The consultation highlighted important benefits of discussion with communities on research design, including providing new perspectives and helping to avoid harm. For example, people living with HIV felt that one of the possible logos could increase stigma within communities. The experience also indicated potential challenges of consultation. In particular, there were contrasting perspectives among the groups, such that the consultation did not provide a clear answer about which logo should be selected. CONCLUSIONS: Our experience adds to current evidence on community engagement by reporting on an area where there is less discussion of community consultation for design of a study logo. The consultation exercise reaffirmed the value of community engagement, but also the difficulty of relying on a brief consultation for decision-making in research design. Further ethical guidance is required on how to negotiate contradictory views during consultations.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6193680/

Record 1018

pmcid: PMC5992494

pmid: 29869582

doi: 10.1108/IJPH-02-2017-0011

title: Creating, reinforcing, and resisting the gender binary: a qualitative study of transgender women's healthcare experiences in sex-segregated jails and prisons.

authors

- White Hughto JM
- Clark KA
- Altice FL
- Reisner SL
- Kershaw TS
- Pachankis JE

journal: International journal of prisoner health

journalAbbr: Int J Prison Health

year: 2018

pubdate: 2018 Jun 11

volume: 14

issue: 2

pages: 69-88

abstract: Purpose Incarcerated transgender women often require healthcare to meet their physical-, mental-, and gender transition-related health needs; however, their healthcare experiences in prisons and jails and interactions with correctional healthcare providers are understudied. The paper aims to discuss these issues. Design/methodology/approach In 2015, 20 transgender women who had been incarcerated in the USA within the past five years participated in semi-structured interviews about their healthcare experiences while incarcerated. Findings Participants described an institutional culture in which their feminine identity was not recognized and the ways in which institutional policies acted as a form of structural stigma that created and reinforced the gender binary and restricted access to healthcare. While some participants attributed healthcare barriers to providers' transgender bias, others attributed barriers to providers' limited knowledge or inexperience

caring for transgender patients. Whether due to institutional (e.g. sex-segregated prisons, biased culture) or interpersonal factors (e.g. biased or inexperienced providers), insufficient access to physical-, mental-, and gender transition-related healthcare negatively impacted participants' health while incarcerated. Research limitations/implications Findings highlight the need for interventions that target multi-level barriers to care in order to improve incarcerated transgender women's access to quality, gender-affirmative healthcare. Originality/value This study provides first-hand accounts of how multi-level forces serve to reinforce the gender binary and negatively impact the health of incarcerated transgender women. Findings also describe incarcerated transgender women's acts of resistance against institutional and interpersonal efforts to maintain the gender binary and present participant-derived recommendations to improve access to gender affirmative healthcare for incarcerated transgender women.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5992494/

Record 1019

pmcid: PMC5938397

pmid: 29765326

doi: 10.3389/fphar.2018.00439

title: Descriptions of Adverse Drug Reactions Are Less Informative in Forums Than in the French Pharmacovigilance Database but Provide More Unexpected Reactions.

authors

- Karapetiantz P
- Bellet F
- Audeh B
- Lardon J
- Leprovost D
- Aboukhamis R
- Morlane-Hondère F
- Grouin C
- Burgun A
- Katsahian S
- Jaulent MC
- Beyens MN
- Lillo-Le Louët A
- Bousquet C

journal: Frontiers in pharmacology

journalAbbr: Front Pharmacol

year: 2018

pubdate: 2018

volume: 9

issue:

pages: 439

abstract: Background: Social media have drawn attention for their potential use in Pharmacovigilance. Recent work showed that it is possible to extract information concerning adverse drug reactions (ADRs) from posts in social media. The main objective of the Vigi4MED project was to evaluate the relevance and quality of the information shared by patients on web forums about drug safety and its potential utility for pharmacovigilance. Methods: After selecting websites of interest, we manually evaluated the relevance of the content of posts for

pharmacovigilance related to six drugs (agomelatine, baclofen, duloxetine, exenatide, strontium ranelate, and tetrazepam). We compared forums to the French Pharmacovigilance Database (FPVD) to (1) evaluate whether they contained relevant information to characterize a pharmacovigilance case report (patient's age and sex; treatment indication, dose and duration; time-to-onset (TTO) and outcome of the ADR, and drug dechallenge and rechallenge) and (2) perform impact analysis (nature, seriousness, unexpectedness, and outcome of the ADR). Results: The cases in the FPVD were significantly more informative than posts in forums for patient description (age, sex), treatment description (dose, duration, TTO), and outcome of the ADR, but the indication for the treatment was more often found in forums. Cases were more often serious in the FPVD than in forums (46% vs. 4%), but forums more often contained an unexpected ADR than the FPVD (24% vs. 17%). Moreover, 197 unexpected ADRs identified in forums were absent from the FPVD and the distribution of the MedDRA System Organ Classes (SOCs) was different between the two data sources. Discussion: This study is the first to evaluate if patients' posts may qualify as potential and informative case reports that should be stored in a pharmacovigilance database in the same way as case reports submitted by health professionals. The posts were less informative (except for the indication) and focused on less serious ADRs than the FPVD cases, but more unexpected ADRs were presented in forums than in the FPVD and their SOCs were different. Thus, web forums should be considered as a secondary, but complementary source for pharmacovigilance.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5938397/

Record 1020

pmcid: PMC5775321

pmid: 29352164

doi: 10.1038/s41598-018-19437-4

title: Determinants of breast size in Asian women.

authors

- Lim LY
- Ho PJ
- Liu J
- Chay WY
- Tan MH
- Hartman M
- Li J

journal: Scientific reports

journalAbbr: Sci Rep

year: 2018

pubdate: 2018 Jan 19

volume: 8

issue: 1

pages: 1201

abstract: Breast size as a risk factor of breast cancer has been studied extensively with inconclusive results. Here we examined the associations between breast size and breast cancer risk factors in 24,353 Asian women aged 50 to 64 years old enrolled in a nationwide mammography screening project conducted between October 1994 and February 1997. Information on demographic and reproductive factors was obtained via a questionnaire. Breast size was ascertained as bust line measured at study recruitment and total breast area measured from a mammogram. The average bust line and total breast area was 91.2 cm and 102.3 cm 2 , respectively. The two breast measurements were moderately correlated (Spearman correlation coefficient = 0.65). Age, BMI, marital and working status were independently associated with bust line and total breast area. In the multivariable analyses, the most pronounced effects were observed for BMI (24.2 cm difference in bust

line and 39.4 cm 2 in breast area comparing women with BMI ≥30 kg/m 2 to BMI <20 kg/m 2). Ethnicity was a positive predictor for total breast area, but not bust line.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5775321/

Record 1021

pmcid: PMC6103999

pmid: 30131065

doi: 10.1186/s13256-018-1772-2

title: Diagnostic difficulties of primary angiosarcoma of the breast: a case report.

authors

- Mahdi Y
- Rouas L
- Malihy A
- Lamalmi N
- Alhamany Z

journal: Journal of medical case reports

journalAbbr: J Med Case Rep

year: 2018

pubdate: 2018 Aug 22

volume: 12

issue: 1

pages: 228

abstract: BACKGROUND: Angiosarcoma of the breast is a rare tumor, which may be primary or secondary to breast surgery or irradiation. It is characterized by polymorphic and nonspecific clinical and radiological features. A pathologist plays a key role in positive and differential diagnosis and in establishing the prognosis: only a histological examination can confirm the diagnosis, and the histologic grade is the most important prognostic factor. In fact, angiosarcomas of the breast constitute a very heterogeneous group and they are classified into three grades based on the degree of differentiation. We will illustrate diagnostic challenges through this new case of primary angiosarcoma of the breast. Microscopic findings were initially interpreted as a benign vascular tumor. We will also discuss the relevant medical literature. CASE PRESENTATION: A 56-year-old Arabian woman presented with a palpable right breast mass that had been enlarging for 2 months, measuring 5 cm, without axillary lymphadenopathy. She had no personal or family history of breast surgery or breast irradiation. A mammography showed no evidence of spiculation. No suspicious calcifications were seen. A needle core biopsy was performed. Microscopic findings were initially interpreted as a benign vascular tumor. However, as the mass measured 5 cm, the diagnosis of angiosarcoma was more appropriate, and mastectomy without axillary dissection was performed. Microscopic examination found mild to moderately scattered pleomorphic cells, and scattered mitotic figures. It also showed papillary formations, solid foci of spindle cells, and hemorrhagic necrosis. The margins of the tumor were infiltrative. The diagnosis of primary intermediately differentiated angiosarcoma of the breast (grade II) was made. No distant metastases were found. Our patient was lost to follow-up and further treatment after mastectomy until she developed local tumor progression 4 months later. CONCLUSIONS: Through this case report, we emphasize the importance of clinicopathological confrontation in angiosarcoma of the breast.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6103999/

Record 1022

pmcid: PMC5758060

pmid: 29322116

doi: 10.5152/ejbh.2017.3416

title: Efficiency of Imaging Modalities in Male Breast Disease: Can Ultrasound Give Additional Information for Assessment of Gynecomastia Evolution?

authors

- Sarıca Ö
- Kahraman AN
- Öztürk E
- Teke M

journal: European journal of breast health

journalAbbr: Eur J Breast Health

year: 2018

pubdate: 2018 Jan

volume: 14

issue: 1

pages: 29-34

abstract: OBJECTIVE: The purpose of this study is to present mammography and ultrasound findings of male breast lesions and to investigate the ability of diagnostic modalities in estimating the evolution of gynecomastia. MATERIALS AND METHODS: Sixty-nine male patients who admitted to Taksim and Bakirkoy Education and Research Hospitals and underwent mammography (MG) and ultrasonography (US) imaging were retrospectively evaluated. Duration of symptoms and mammographic types of gynecomastia according to Appelbaum's classifications were evaluated, besides the sonographic findings in mammographic types of gynecomastia. RESULTS: The distribution of 69 cases were as follows: gynecomastia 47 (68.11%), pseudogynecomastia 6 (8.69%) primary breast carcinoma 7 (10.14%), metastatic carcinoma 1 (1.4%), epidermal inclusion cyst 2 (2.8%), abscess 2 (2.8%), lipoma 2 (2.8%), pyogenic granuloma 1 (1.4%), and granulomatous lobular mastitis 1 (1.4%). Gynecomastia patients who had symptoms less than 1 year had nodular gynecomastia (34.6%) as opposed to dendritic gynecomastia (61.5%) ($p<0.01$) based on mammography results according to Appelbaum's classifications. In patients having symptoms for 1 to 2 years, diffuse gynecomastia (70%) had a higher rate than the dendritic type (20%). Patients having the symptoms more than 2 years had diffuse gynecomastia (57.1%) while 42.9% had dendritic gynecomastia ($p<0.001$). With sonographic examination patients who had symptoms less than 1 year had higher rates of dendritic gynecomastia (92.3%) than noduler type (1.9 %). Patients having symptoms for 1 to 2 years had more dentritic gynecomastia (70%) than diffuse type (30%). Patients having symptoms more than 2 years had diffuse gynecomastia (57.1%) comparable to dendritic gynecomastia (42.9 %). CONCLUSION: Diagnostic imaging modalities are efficient tools for estimation of gynecomastia evolution as well as the diagnosis of other male breast diseases. There seems to be an incongruity between duration of clinical complaints and diagnostic imaging classification of gynecomastia. The use of these high resolution US findings may demonstrate an early phase fibrosis especially in patients visualized by mammography as with nodular phase.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5758060/

Record 1023

pmcid: PMC7098080

pmid: 29372930

doi: 10.1002/14651858.MR000039.pub2

title: Eliciting adverse effects data from participants in clinical trials.

authors

- Allen EN

- Chandler CI
- Mandimika N
- Leisegang C
- Barnes K

journal: The Cochrane database of systematic reviews

journalAbbr: Cochrane Database Syst Rev

year: 2018

pubdate: 2018 Jan 16

volume: 1

issue: 1

pages: MR000039

abstract: BACKGROUND: Analysis of drug safety in clinical trials involves assessing adverse events (AEs) individually or by aggregate statistical synthesis to provide evidence of likely adverse drug reactions (ADR). While some AEs may be ascertained from physical examinations or tests, there is great reliance on reports from participants to detect subjective symptoms, where he/she is often the only source of information. There is no consensus on how these reports should be elicited, although it is known that questioning methods influence the extent and nature of data detected. This leaves room for measurement error and undermines comparisons between studies and pooled analyses. This review investigated comparisons of methods used in trials to elicit participant-reported AEs. This should contribute to knowledge about the methodological challenges and possible solutions for achieving better, or more consistent, AE ascertainment in trials. OBJECTIVES: To systematically review the research that has compared methods used within clinical drug trials (or methods that would be specific for such trials) to elicit information about AEs defined in the protocol or in the planning for the trial. SEARCH METHODS: Databases (searched to March 2015 unless indicated otherwise) included: Embase; MEDLINE; MEDLINE in Process and Other Non-Indexed Citations; Cochrane Methodology Register (July 2012); Cochrane Central Register of Controlled Trials (February 2015); Cochrane Database of Systematic Reviews; Database of Abstracts of Reviews of Effects (January 2015); Health Technology Assessment database (January 2015); CINAHL; CAB Abstracts; BIOSIS (July 2013); Science Citation Index; Social Science Citation Index; Conference Proceedings Citation Index - Science. The search used thesaurus headings and synonyms for the following concepts: (A): Adverse events AND measurement; (B): Participants AND elicitation (also other synonyms for extraction of information about adverse effects from people); (C): Participants AND checklists (also other synonyms as for B). Pragmatic ways were used to limit the results whilst trying to maintain sensitivity. There were no date or sample size restrictions but only reports published in English were included fully, because of resource constraints as regards translation. SELECTION CRITERIA: Two types of studies were included: drug trials comparing two or more methods within- or between-participants to elicit participant-reported AEs, and research studies performed outside the context of a trial to compare methods which could be used in trials (evidenced by reference to such applicability). Primary outcome data included AEs elicited from participants taking part in any such clinical trial. We included any participant-reported data relevant for an assessment of drug-related harm, using the original authors' terminology (and definition, where available), with comment on whether the data were likely to be treatment-emergent AEs or not. DATA COLLECTION AND ANALYSIS: Titles and abstracts were independently reviewed for eligibility. Full texts of potentially eligible citations were independently reviewed for final eligibility. Relevant data were extracted and subjected to a 100% check. Disagreements were resolved by discussion, involving a third author. The risk of bias was independently assessed by two authors. The Cochrane 'Risk of bias' tool was used for reports comparing outcomes between participants, while for within-participant comparisons, each study was critically evaluated in terms of potential impact of the design and conduct on findings using the framework of selection, performance, detection, attrition, reporting, and other biases. An attempt was made to contact authors to retrieve protocols or specific relevant missing information. Reports were not excluded on the basis of quality unless data for outcomes were impossible to compare (e.g. where denominators differed). A narrative synthesis was conducted because differences in study design and presentation meant that a quantitative meta-analysis was not possible. MAIN RESULTS: The 33 eligible studies largely compared open questions with checklist-type questions or rating scales. Two included participant interviews. Despite different designs, populations and details of questioning methods, the narrative review showed that more specific questioning of participants led to more AEs detected

compared to a more general enquiry. A subset of six studies suggested that more severe, bothersome, or otherwise clinically relevant AEs were reported when an initial open enquiry was used, while some less severe, bothersome, or clinically relevant AEs were only reported with a subsequent specific enquiry. However, two studies showed that quite severe or debilitating AEs were only detected by an interview, while other studies did not find a difference in the nature of AEs between elicitation methods. No conclusions could be made regarding the impact of question method on the ability to detect a statistically significant difference between study groups. There was no common statistical rubric, but we were able to represent some effect measures as a risk ratio of the proportion of participants with at least one AE. This showed a lower level of reporting for open questions (O) compared to checklists (CL), with a range for the risk ratios of 0.12 to 0.64. AUTHORS' CONCLUSIONS: This review supports concerns that methods to elicit participant-reported AEs influence the detection of these data. There was a risk for under-detection of AEs in studies using a more general elicitation method compared to those using a comprehensive method. These AEs may be important from a clinical perspective or for patients. This under-detection could compromise ability to pool AE data. However, the impact on the nature of the AE detected by different methods is unclear. The wide variety and low quality of methods to compare elicitation strategies limited this review. Future studies would be improved by using and reporting clear definitions and terminology for AEs (and other important variables), frequency and time period over which they were ascertained, how they were graded, assessed for a relationship to the study drug, coded, and tabulated/reported. While the many potential AE endpoints in a trial may preclude the development of general AE patient-reported outcome measurement instruments, much could also be learnt from how these employ both quantitative and qualitative methods to better understand data elicited. Any chosen questioning method needs to be feasible for use by both staff and participants.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7098080/

Record 1024

pmcid: PMC5869434

pmid: 29506328

doi: 10.5999/aps.2017.01263

title: Endoscopic transaxillary prepectoral conversion for submuscular breast implants.

authors

- Park SH
- Sim HB

journal: Archives of plastic surgery

journalAbbr: Arch Plast Surg

year: 2018

pubdate: 2018 Mar

volume: 45

issue: 2

pages: 158-164

abstract: BACKGROUND: During breast augmentation, the transaxillary approach provides the advantage of allowing the mammary prosthesis to be placed through incisions that are remote from the breast itself, thereby reducing the visibility of postoperative scars. For patients experiencing capsular contracture who do not want additional scars, the previous transaxillary scar can be used for site change and implant exchange. METHODS: This study analyzed 17 patients (34 breasts) with submuscular breast implants with grade III-IV capsular contracture who received treatment from 2010 to 2015. The mean age of the patients was 29 years (range, 20-38 years). The inclusion criterion was a pinch test of more than 3 cm at the upper pole of the breast. Previous axillary scars were used to expose the pectoralis fascia, and submuscular breast implants were removed carefully. The dissection underneath the pectoralis fascia was performed with endoscopic assistance, using electrocautery under direct visualization. RESULTS: The mean follow-up period was 14 months (range, 6-24 months). The entire dissection plane was changed from the submuscular plane to the subfascial plane.

Round textured gel implants were used, with a mean implant size of 220 mL (range, 160-300 mL). Two patients developed grade II capsular contracture. There were no cases of malposition or asymmetry. Three patients complained of minor implant palpability. None of the patients required additional surgery. CONCLUSIONS: Endoscopic subfascial conversion may be an effective technique for treating capsular contracture and avoiding scarring of the breast in selected patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5869434/

Record 1025

pmcid: PMC6004900

pmid: 29733030

doi: 10.3233/JRS-180744

title: Enduring sexual dysfunction after treatment with antidepressants, 5α-reductase inhibitors and isotretinoin: 300 cases.

authors

- Healy D
- Le Noury J
- Mangin D

journal: The International journal of risk & safety in medicine

journalAbbr: Int J Risk Saf Med

year: 2018

pubdate: 2018

volume: 29

issue: 3-4

pages: 125-134

abstract: OBJECTIVE: To investigate clinical reports of post-SSRI sexual dysfunction (PSSD), post-finasteride syndrome (PFS) and enduring sexual dysfunction following isotretinoin. METHODS: Data from RxISK.org, a global adverse event reporting website, have been used to establish the clinical features, demographic details and clinical trajectories of syndromes of persistent sexual difficulties following three superficially different treatment modalities.RESULTSWe report on 300 cases of enduring sexual dysfunction from 37 countries following 14 different drugs comprised of serotonin reuptake inhibiting antidepressants, 5α-reductase inhibitors and isotretinoin. While reports of certain issues were unique to the antidepressants, such as the onset of premature ejaculation and persistent genital arousal disorder (PGAD), there was also a significant overlap in symptom profile between the drug groups, with common features including genital anaesthesia, pleasureless or weak orgasm, loss of libido and impotence. Secondary consequences included relationship breakdown and impaired quality of life.CONCLUSIONSThese data point to a legacy syndrome or syndromes comprising a range of disturbances to sexual function. More detailed studies will require developments in coding systems that recognise the condition(s). Further exploration of these tardive sexual syndromes may yield greater understanding of tardive syndromes in general.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6004900/

Record 1026

pmcid: PMC5773955

pmid: 29239131

doi: 10.1002/cam4.1273

title: Finasteride treatment and male breast cancer: a register-based cohort study in four Nordic countries.

authors

- Meijer M
- Thygesen LC
- Green A
- Emneus M
- Brasso K
- Iversen P
- Pukkala E
- Bolin K
- Stavem K
- Ersbøll AK

journal: Cancer medicine

journalAbbr: Cancer Med

year: 2018

pubdate: 2018 Jan

volume: 7

issue: 1

pages: 254-260

abstract: A potential link has been suggested between dispensed finasteride and increased risk of male breast cancer (MBC). Due to the rare occurrence of MBC, it remains to be established if such a relationship exists. The purpose of this study was to combine nationwide registers in four countries to assess the potential association between dispensed finasteride and MBC. A cohort of all males with dispensed finasteride in Denmark, Finland, Norway, and Sweden (1,365,088 person years) was followed up for up to 15 years for breast cancer, and compared to a cohort of males unexposed to finasteride. Individual-level register data included country, dates of dispensed finasteride, MBC diagnosis, and death. Incidence rate ratios (IRRs) were estimated using a generalized linear model with a Poisson distribution. An increased risk of MBC was found among finasteride users (IRR = 1.44, 95% confidence interval [95% CI] = 1.11-1.88) compared to nonusers. The IRR increased to 1.60 (95% CI = 1.20-2.13) when users in Norway and Sweden with short follow-up time were excluded. The highest IRR was seen among men with medium duration of dispensed finasteride, medium accumulated consumption of finasteride, and among men with first dispensed finasteride prescription 1-3 years prior to diagnosis. The analyses suggested possible ascertainment bias and did not support a clear relationship between dispensed finasteride and MBC. In conclusion, a significant association between dispensed finasteride and MBC was identified. However, due to limited data for adjustment of potential confounding and surveillance bias in the present study, further research is needed to confirm these results.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5773955/

Record 1027

pmcid: PMC6174656

pmid:

doi:

title: Friday, September 28, 2018, 09:00-10:00 hrs, E-Poster designated area: E Poster Session – 01: Miscellaneous Disorders.

authors

journal: Annals of Indian Academy of Neurology

journalAbbr: Ann Indian Acad Neurol

year: 2018

pubdate: 2018 Sep

volume: 21

issue: Suppl 2

pages: S72-S114

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6174656/

Record 1028

pmcid: PMC5878311

pmid: 29592998

doi: 10.1136/bcr-2017-224009

title: Gestational gigantomastia with spontaneous resolution in an Indian woman.

authors

- Dharini
- Venkataram T
- Raghuprakash S

journal: BMJ case reports

journalAbbr: BMJ Case Rep

year: 2018

pubdate: 2018 Mar 28

volume: 2018

issue:

pages:

abstract: We present the case of a 20-year-old woman, primigravida with 24 weeks of amenorrhoea due to pregnancy. She came with symptoms of rapidly increasing size of both the breasts since 8 weeks. On examination, both breasts were grossly enlarged, right more than left, with the level of the nipples reaching beyond the level of the umbilicus. Breast circumference was measured to be 72 cm on the right, and left breast was 66 cm. No palpable lumps in both breasts. No axillary lymphadenopathy noted. Hormonal studies were found to be within the normal range. The pregnancy was terminated due to pre-eclampsia. The patient opted for regular follow-up and observation. While regularly following up, the size of both breasts was observed to gradually reduce to a circumference of 40 cm on the right and 30 cm on the left.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5878311/

Record 1029

pmcid: PMC6099052

pmid: 30147887

doi: 10.1002/ccr3.1565

title: Gynecomastia after euthyroidism restoration in a patient with type 1 diabetes and Graves' disease.

authors

- Calcaterra V
- Clerici E
- Ceolin V

- Regalbuto C
- Larizza D

journal: Clinical case reports

journalAbbr: Clin Case Rep

year: 2018

pubdate: 2018 Aug

volume: 6

issue: 8

pages: 1481-1484

abstract: In patients with autoimmune disease, gynecomastia should not be considered as 1 of the first signs of hyperthyroidism, rather it is a breast pathology that can be present even when euthyroidism restoration is achieved. It is unknown whether the autoimmune nature of thyroid disorders or simply the hyperthyroidism effects breast changes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6099052/

Record 1030

pmcid: PMC6324867

pmid: 30595602

doi: 10.12659/AJCR.913456

title: Imaging and Clinical Features of an Unusual Unilateral Breast Enlargement Diagnosed as Fibrocystic Change: A Case Report.

authors

- Kim SJ
- Kim WG

journal: The American journal of case reports

journalAbbr: Am J Case Rep

year: 2018

pubdate: 2018 Dec 31

volume: 19

issue:

pages: 1550-1555

abstract: BACKGROUND Breasts are assumed to be symmetrical bilaterally, and abnormal findings on breast imaging are largely based on such an assumption. Clinically noticeable breast asymmetry beyond that of normal range is rarely encountered. CASE REPORT A 38-year-old female presented with unilateral enlargement of her left breast for 3 months and complained of polymenorrhea twice a month. Mammography and ultrasonography revealed that the left breast had a larger volume of fibroglandular tissue than the right breast, without accompanying signs of malignancy or abnormality. Magnetic resonance imaging demonstrated unilateral, diffuse, stippled enhancement in the left breast, which was located peripherally in the early phase and propagated centrally in the delayed phase with a persistent kinetic pattern. Ultrasonography-guided core needle biopsy was performed for the left breast, leading to a pathological diagnosis of fibrocystic change. The condition could be presumably due to a different response of the breasts to imbalance in endogenous hormones. CONCLUSIONS Based on our findings, we believe that radiologists should consider that the breast has a unique dynamic physiology and that features on breast imaging can be affected by hormonal alteration.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6324867/

Record 1031

pmcid: PMC5768507

pmid: 29353994

doi: 10.3348/kjr.2018.19.1.5

title: Imaging Features of Inflammatory Breast Disorders: A Pictorial Essay.

authors

- Leong PW
- Chotai NC
- Kulkarni S

journal: Korean journal of radiology

journalAbbr: Korean J Radiol

year: 2018

pubdate: 2018 Jan-Feb

volume: 19

issue: 1

pages: 5-14

abstract: Inflammatory breast disorders include a wide array of underlying causes, ranging from common benign infection, non-infectious inflammation and inflammation resulting from underlying breast malignancy. Because it is at times difficult to distinguish mastitis and breast cancer based on clinical features, awareness of detailed imaging features may be helpful for better management of inflammatory breast disorders. Therefore, this pictorial essay intends to demonstrate radiologic findings of a variety of inflammatory breast disorders, using selected cases with mammography, ultrasound and magnetic resonance images.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5768507/

Record 1032

pmcid: PMC6104939

pmid: 30133462

doi: 10.1371/journal.pone.0201906

title: Increased final adult height by gonadotropin-releasing hormone agonist in girls with idiopathic central precocious puberty.

authors

- Lee HS
- Yoon JS
- Park KJ
- Hwang JS

journal: PloS one

journalAbbr: PLoS One

year: 2018

pubdate: 2018

volume: 13

issue: 8

pages: e0201906

abstract: OBJECTIVE: Gonadotropin-releasing hormone agonists (GnRHa) are the treatment of choice for central precocious puberty (CPP) and have been widely used for several decades. We determined the effect of GnRHa treatment on the auxological outcomes of girls with idiopathic CPP. METHODS: This study included 84 girls treated monthly with depot leuprolide acetate who had reached adult height. We compared their final adult height (FAH) with their initial predicted adult height (PAH). We performed a multivariate analysis of the factors associated with FAH on all girls diagnosed with CPP. RESULTS: We performed the final evaluations at a mean age of 14.1 ± 0.8 years after a mean treatment duration of 2.98 ± 0.73 years (ranging from 1.5-4.8 years). Menarche had occurred at 12.6 ± 0.6 years of age, which was 16.5 ± 6.1 months after discontinuation of GnRHa therapy. Mean FAH was 160.1 ± 5.0 cm, which was significantly higher than the initial PAH (156.1 ± 5.7 cm; $P < 0.001$). To investigate whether growth outcomes were influenced by the age at initial treatment, we divided all patients into two groups, those treated between 6 and 8 years (n = 23) and those treated after 8 years (n = 61); no significant differences were observed in FAH between the two groups. FAH was significantly and positively correlated with the height standard deviation score (SDS) at the end of treatment and with the target height, whereas the difference between bone age and chronological age at the start and end of treatment was negatively correlated with FAH. CONCLUSION: FAH was significantly higher than the initial PAH in girls with CPP who were treated with GnRHa. Also, GnRHa treatment was still effective even after 8 years of age in girls with CPP.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6104939/

Record 1033

pmcid: PMC5782395

pmid: 29361985

doi: 10.1186/s13058-017-0930-6

title: Inherited factors contribute to an inverse association between preeclampsia and breast cancer.

authors

- Yang H
- He W
- Eriksson M
- Li J
- Holowko N
- Chiesa F
- Hall P
- Czene K

journal: Breast cancer research : BCR

journalAbbr: Breast Cancer Res

year: 2018

pubdate: 2018 Jan 23

volume: 20

issue: 1

pages: 6

abstract: BACKGROUND: Preeclampsia is frequently linked to reduced breast cancer risk. However, little is known regarding the underlying genetic association and the association between preeclampsia and mammographic density. METHODS: This study estimates the incidence rate ratios (IRRs) of breast cancer in patients with preeclampsia, when compared to women without preeclampsia, using Poisson regression models in two cohorts of pregnant women: a Swedish nationwide cohort (n = 1,337,934, 1973-2011) and the Karolinska Mammography Project for Risk Prediction of Breast Cancer (KARMA, n = 55,044, 1958-2015). To identify the

genetic association between preeclampsia and breast cancer, we used logistic regression models to calculate the odds ratios (ORs) of preeclampsia in sisters of breast cancer patients, and in women with different percentiles of breast cancer polygenic risk scores (PRS). Linear regression models were used to estimate the mammographic density by preeclampsia status in the KARMA cohort. RESULTS: A decreased risk of breast cancer was observed among patients with preeclampsia in both the nationwide (IRR = 0.90, 95% CI = 0.85; 0.96) and KARMA cohorts (IRR = 0.75, 95% CI = 0.61; 0.93). Women with high breast cancer PRS and sisters of breast cancer patients had a lower risk of preeclampsia (OR = 0.89, 95% CI = 0.83; 0.96). Mammographic density was lower in women with preeclampsia compared to women without preeclampsia (-2.04%, 95% CI = -2.65; -1.43). Additionally, among sisters in the KARMA cohort (N = 3500), density was lower in sisters of patients with preeclampsia compared to sisters of women without preeclampsia (-2.76%, 95% CI = -4.96; -0.56). CONCLUSION: Preeclampsia is associated with reduced risk of breast cancer and mammographic density. Inherited factors contribute to this inverse association.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5782395/

Record 1034

pmcid: PMC6039200

pmid: 29637739

doi: 10.1111/cts.12545

title: Initial Clinical Experience of RP5063 Following Single Doses in Normal Healthy Volunteers and Multiple Doses in Patients with Stable Schizophrenia.

authors

- Cantillon M
- Ings R
- Bhat L

journal: Clinical and translational science

journalAbbr: Clin Transl Sci

year: 2018

pubdate: 2018 Jul

volume: 11

issue: 4

pages: 387-396

abstract: RP5063 is a multimodal dopamine (D)-serotonin (5-HT) stabilizer with a high affinity for D 2/3/4 and 5-HT 1A/2A/2B/7 receptors and moderate affinity for the serotonin transporter. Single-dose (10 and 15 mg fasting, 15 mg fed) safety in healthy volunteers and multiple-dose (10, 20, 50, and 100 mg fed, 10 days) safety and pharmacodynamics in patients with stable schizophrenia were defined in two phase I studies. In the single-dose study, 32 treatment-emergent adverse events (TEAEs) were observed. Orthostatic hypotension (n = 6), nausea (n = 5), and dizziness (n = 4) were the most common. One serious adverse event (SAE), seen in a patient who should not have been in the study due to a history of seizures, involved brief seizure-like symptoms. In the multiple-dose study, 75 TEAEs were reported. Akathisia (n = 20) and somnolence (n = 14) were the most frequent. No clinically significant changes were seen in glucose or prolactin levels, lipid profiles, weight, or electrocardiographic recordings. In both studies, all TEAEs resolved and none led to withdrawal from the study or death. A pharmacodynamic evaluation reflected significant improvements with RP5063 ($P < 0.05$) over placebo in an analysis of patients with a baseline Positive and Negative Syndrome Scale (PANSS) score ≥50 for positive subscale scores. Improvements of the Trail Making A and Trail Making B test results were observed for patients treated in the 50 mg dose group for days 5, 10, and 16. These findings indicate that RP5063 is well-tolerated up to 100 mg and displays promising preliminary clinical behavioral and cognition activity signals in patients with stable disease over a 10-day period.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6039200/

Record 1035

pmcid: PMC6085687

pmid: 30092829

doi: 10.1186/s13058-018-1026-7

title: Long-term exposure to insulin and volumetric mammographic density: observational and genetic associations in the Karma study.

authors

- Borgquist S
- Rosendahl AH
- Czene K
- Bhoo-Pathy N
- Dorkhan M
- Hall P
- Brand JS

journal: Breast cancer research : BCR

journalAbbr: Breast Cancer Res

year: 2018

pubdate: 2018 Aug 9

volume: 20

issue: 1

pages: 93

abstract: BACKGROUND: Long-term insulin exposure has been implicated in breast cancer etiology, but epidemiological evidence remains inconclusive. The aims of this study were to investigate the association of insulin therapy with mammographic density (MD) as an intermediate phenotype for breast cancer and to assess associations with long-term elevated circulating insulin levels using a genetic score comprising 18 insulin-associated variants. METHODS: We used data from the KARolinska MAmmography (Karma) project, a Swedish mammography screening cohort. Insulin-treated patients with type 1 (T1D, n = 122) and type 2 (T2D, n = 237) diabetes were identified through linkage with the Prescribed Drug Register and age-matched to 1771 women without diabetes. We assessed associations with treatment duration and insulin glargine use, and we further examined MD differences using non-insulin-treated T2D patients as an active comparator. MD was measured using a fully automated volumetric method, and analyses were adjusted for multiple potential confounders. Associations with the insulin genetic score were assessed in 9437 study participants without diabetes. RESULTS: Compared with age-matched women without diabetes, insulin-treated T1D patients had greater percent dense (8.7% vs. 11.4%) and absolute dense volumes (59.7 vs. 64.7 cm 3), and a smaller absolute nondense volume (615 vs. 491 cm 3). Similar associations were observed for insulin-treated T2D, and estimates were not materially different in analyses comparing insulin-treated T2D patients with T2D patients receiving noninsulin glucose-lowering medication. In both T1D and T2D, the magnitude of the association with the absolute dense volume was highest for long-term insulin therapy (≥ 5 years) and the long-acting insulin analog glargine. No consistent evidence of differential associations by insulin treatment duration or type was found for percent dense and absolute nondense volumes. Genetically predicted insulin levels were positively associated with percent dense and absolute dense volumes, but not with the absolute nondense volume (percentage difference [95% CI] per 1-SD increase in insulin genetic score = 0.8 [0.0; 1.6], 0.9 [0.1; 1.8], and 0.1 [- 0.8; 0.9], respectively). CONCLUSIONS: The consistency in direction of association for insulin treatment and the insulin genetic score with the absolute dense volume suggest a causal influence of long-term increased insulin exposure on mammographic dense breast tissue.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6085687/

Record 1036

pmcid: PMC7061539

pmid: 32158801

doi: 10.1016/j.jpra.2017.10.007

title: Low rate of capsular contracture in a series of 214 consecutive primary and revision breast augmentations using microtextured implants.

authors

- Tanner B

journal: JPRAS open

journalAbbr: JPRAS Open

year: 2018

pubdate: 2018 Mar

volume: 15

issue:

pages: 66-73

abstract: Capsular contracture is the most common major complication after implant-based breast augmentation. The aetiology of capsular contracture is multifactorial. The author reports a retrospective personal series of patients managed over a seven-year period with a nearly unchanged surgical strategy implementing most of known measures for capsular contracture prevention. A microtextured silicone gel-filled implant from a single manufacturer was used in all cases. There were 214 consecutive patients (126 primary augmentations and 88 revision augmentations) operated on over the study period. Mean age of the population was 40.0 years, and mean BMI was 22.0 kg/m 2 . Of the patients in the revision cohort, 44.3% were reoperated on because of previous capsular contracture. Average follow-up was 20.2 months. There was a 0% capsular contracture rate in the primary augmentation cohort and a 3.4% capsular contracture rate in the revision cohort. At last follow-up, 91.2% of breasts received a Baker I grading. Although the follow-up was relatively short, this rate of capsular contracture would still be considered very low. Determining the reason for such a low rate of capsular contracture on Multivariate Analyses would be difficult due to the potential myriad of confounding variables. However, given the constancy of the technique and implant type employed by a single surgeon, the author is of the opinion that the microtexturing topography on the implant surfaceused in this series contributed to the low rate of capsular contracture formation. However, this would need to be tested in arandomized controlled trial comparing microtextured devices with implants that have macrotextured surfaces.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7061539/

Record 1037

pmcid: PMC5758064

pmid: 29322112

doi: 10.5152/ejbh.2017.3841

title: Managing Male Mammary Maladies.

authors

- Fentiman IS

journal: European journal of breast health

journalAbbr: Eur J Breast Health

year: 2018

pubdate: 2018 Jan

volume: 14

issue: 1

pages: 5-9

abstract: This review examines the symptoms, need for referral and management of the benign breast conditions which afflict males, together with the steps that are necessary to exclude or confirm male breast cancer. The most common complaint is gynaecomastia, either true or pseudo, and the majority of these cases need reassurance without over-investigation. Drugs that induce breast enlargement are described in order that, when possible, a medication switch can be made. Men receiving endocrine therapy for prostate cancer may develop painful gynaecomastia and this can be relieved with tamoxifen. All men with breast cancer need mammography as part of their work-up but this should not be used as a screening technique for symptomatic males. Because of lack of lobular development, both cysts and fibroadenomas are very rare in men; but those with nipple discharge need referral and investigation as some will have underlying malignancy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5758064/

Record 1038

pmcid: PMC6211884

pmid: 30335007

doi: 10.1097/MD.0000000000012879

title: Nonalcoholic fatty liver disease with prolactin-secreting pituitary adenoma in an adolescent: A case report.

authors

- Takaki Y
- Mizuochi T
- Nishioka J
- Eda K
- Yatsuga S
- Yamashita Y

journal: Medicine

journalAbbr: Medicine (Baltimore)

year: 2018

pubdate: 2018 Oct

volume: 97

issue: 42

pages: e12879

abstract: RATIONALE: Nonalcoholic fatty liver disease (NAFLD), among the commonest chronic liver disorders in children and adolescents, is considered a reflection of the current obesity epidemic in children and adults. This liver disease has been linked with various metabolic disorders, but not with prolactinoma (PRLoma). PATIENT CONCERNS: A 13-year-old Japanese girl manifested obesity, serum transaminase and γ-glutamyltransferase elevations, and amenorrhea. Abdominal ultrasonography showed fatty liver. Her serum prolactin concentration was elevated, and cranial magnetic resonance imaging showed a pituitary mass consistent with macroadenoma. DIAGNOSES: NAFLD and PRLoma. INTERVENTIONS AND OUTCOMES: After the patient's NAFLD failed to respond to diet and exercise, cabergoline treatment of the PRLoma decreased body weight, serum transaminase and γ-glutamyltransferase elevations, and ultrasonographic fatty liver grade as the tumor became smaller. LESSONS: Physicians should consider the possibility of PRLoma when diet and exercise fail to improve fatty liver disease in a patient with endocrine symptoms such as amenorrhea.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6211884/

Record 1039

pmcid: PMC5771532

pmid: 29064335

doi: 10.1089/jwh.2017.6385

title: Overall Safety of Ospemifene in Postmenopausal Women from Placebo-Controlled Phase 2 and 3 Trials.

authors

- Simon JA
- Altomare C
- Cort S
- Jiang W
- Pinkerton JV

journal: Journal of women's health (2002)

journalAbbr: J Womens Health (Larchmt)

year: 2018

pubdate: 2018 Jan

volume: 27

issue: 1

pages: 14-23

abstract: OBJECTIVE: To evaluate the safety of daily oral ospemifene 60 mg, estrogen agonist/antagonist, used to treat moderate-to-severe dyspareunia due to postmenopausal vulvovaginal atrophy, which is part of genitourinary syndrome of menopause. METHODS: Post hoc analysis of safety data (treatment-emergent adverse events [TEAEs]) pooled from six phase 2 and 3 randomized, double-blind, multicenter placebo-controlled studies, evaluating the effects of ospemifene 60 mg on the breast, cardiovascular system, and bone in postmenopausal women. RESULTS: At least one TEAE was reported by 67.6% (840/1242) and 54.1% (518/958) of women taking ospemifene 60 mg and placebo, respectively. Most TEAEs were mild or moderate and occurred within 4 to 12 weeks. The most commonly reported TEAEs with ospemifene were hot flush (8.5% vs. 3.3% for placebo) and urinary tract infection (6.5% vs. 4.8%). Discontinuation due to TEAEs was 7.6% with ospemifene and 3.8% with placebo. Most women discontinued treatment due to adverse events (AEs): hot flushes, muscle spasms, headache, and vaginal discharge. Serious AEs occurred infrequently (ospemifene, 2.6%; placebo, 1.8%); most were not considered related to treatment. Breast cancer and other breast-related TEAE incidences were comparable between ospemifene (2.5%) and placebo (2.2%), and cardiovascular TEAE incidence, including deep vein thrombosis, was low with ospemifene (0.3%) and placebo (0.1%). CONCLUSION: No unexpected safety signals were reported, and discontinuation due to TEAEs was low, with use of ospemifene 60 mg versus placebo in six phase 2 and 3 trials, suggesting a lack of detrimental effects on the breast, bone, and cardiovascular health of postmenopausal women when ospemifene is used to effectively treat moderate-to-severe postmenopausal dyspareunia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5771532/

Record 1040

pmcid: PMC5811845

pmid: 29479420

doi: 10.1093/jscr/rjy024

title: Preoperative radiation in large angiomatosis of the breast, attempting breast conserving surgery: multidisciplinary approach.

authors

- Chulakadabba A
- Denariyakoon S
- Chakkabat P
- Shotelersuk K
- Sampatanukul P
- Boonjunwetwat D
- Chatamra K

journal: Journal of surgical case reports

journalAbbr: J Surg Case Rep

year: 2018

pubdate: 2018 Feb

volume: 2018

issue: 2

pages: rjy024

abstract: Angiomatosis of the breast is very rare. The presentations are including breast mass, skin discoloration and breast enlargement that mimic to angiosarcoma. The imaging could suggest non-specific vascular tumors. The histology should be obtained for the certain diagnosis. Surgical excision is the standard treatment. To our knowledge, this is the first case that preoperative radiation is given. Currently, radiation is occasionally used in benign condition. This case shows the successful result of preoperative radiation for achieving breast conserving surgery in large angiomatosis of the breast.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5811845/

Record 1041

pmcid: PMC6034137

pmid: 29988973

doi: 10.1016/j.radcr.2018.04.009

title: Primary lymphoma of the breast: A case series.

authors

- Wadhwa A
- Seneboutarath K

journal: Radiology case reports

journalAbbr: Radiol Case Rep

year: 2018

pubdate: 2018 Aug

volume: 13

issue: 4

pages: 815-821

abstract: Primary breast lymphoma has been defined as localized involvement of one or both breasts with or without ipsilateral axillary nodal involvement, usually as a rare manifestation of extranodal non-Hodgkin's lymphoma. We describe three cases of this uncommon disease seen at our institution. Two of these cases presented as palpable breast lumps and one was screening detected. None of the patients had a prior history of lymphoma; only one reported B symptoms night sweats and weight loss. Diagnosis was established on ultrasound-guided core biopsy and no evidence of metastatic disease was identified. Even though the imaging

features are not specific to this diagnosis, the radiologists should be aware of the clinical and imaging presentation of this rare malignancy to recommend appropriate management and establish radiologic-pathologic concordance.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6034137/

Record 1042

pmcid: PMC6837776

pmid: 31754517

doi: 10.4102/sajr.v22i2.1366

title: Pseudoangiomatous stromal hyperplasia: Presentation and management - a clinical perspective.

authors

- Smilg P

journal: SA journal of radiology

journalAbbr: SA J Radiol

year: 2018

pubdate: 2018

volume: 22

issue: 2

pages: 1366

abstract: Pseudoangiomatous stromal hyperplasia (PASH) is a benign breast condition of collagen proliferation. In this article, four cases are presented in series to illustrate the varying clinical presentations of PASH at mammography and sonography, as well as the vastly differing age groups that can be affected. A literature review of the aetiology, pathology and management of PASH is included to provide a comprehensive but succinct overview of the condition, what it is and how to recognise and manage it.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6837776/

Record 1043

pmcid: PMC6280256

pmid: 30584530

doi: 10.1155/2018/1250721

title: Rapid-Onset Obesity with Hypoventilation, Hypothalamic, Autonomic Dysregulation, and Neuroendocrine Tumors (ROHHADNET) Syndrome: A Systematic Review.

authors

- Lee JM
- Shin J
- Kim S
- Gee HY
- Lee JS
- Cha DH
- Rim JH
- Park SJ
- Kim JH
- Uçar A

- Kronbichler A
- Lee KH
- Shin JI

journal: BioMed research international

journalAbbr: Biomed Res Int

year: 2018

pubdate: 2018

volume: 2018

issue:

pages: 1250721

abstract: BACKGROUND AND AIM: ROHHADNET (rapid-onset obesity with hypoventilation, hypothalamic, autonomic dysregulation, neuroendocrine tumor) syndrome is a rare disease with grave outcome. Although early recognition is essential, prompt diagnosis may be challenging due to its extreme rarity. This study aimed to systematically review its clinical manifestation and to identify genetic causes. MATERIALS AND METHODS: We firstly conducted a systematic review on ROHHAD/NET. Electronic databases were searched using related terms. We secondly performed whole exome sequencing (WES) and examined copy number variation (CNV) in two patients to identify genetic causes. RESULTS: In total, 46 eligible studies including 158 patients were included. There were 36 case reports available for individual patient data (IPD; 48 patients, 23 ROHHAD, and 25 ROHHADNET) and 10 case series available for aggregate patient data (APD; 110 patients, 71 ROHHAD, and 39 ROHHADNET). The median age at onset calculated from IPD was 4 years. Gender information was available in 100 patients (40 from IPD and 60 from APD) in which 65 females and 35 males were showing female preponderance. Earliest manifestation was rapid obesity, followed by hypothalamic symptoms. Most common types of neuroendocrine tumors were ganglioneuromas. Patients frequently had dysnatremia and hyperprolactinemia. Two patients were available for WES. Rare variants were identified in PIK3R3 , SPTBN5, and PCF11 in one patient and SRMS , ZNF83, and KMT2B in another patient, respectively. However, there was no surviving variant shared by the two patients after filtering. CONCLUSIONS: This study systematically reviewed the phenotype of ROHHAD/NET aiming to help early recognition and reducing morbidity. The link of variants identified in the present WES requires further investigation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6280256/

Record 1044

pmcid: PMC5869046

pmid: 29588915

doi: 10.1055/s-0038-1637000

title: Recurrent 8-Year Ongoing Unilateral Breast Seroma Formation after PIP Implant Removal-A Case Report and Review of the Literature.

authors

- Kästner S
- Paprottka FJ
- Gonser P
- López MV
- Kaye KO

journal: Surgery journal (New York, N.Y.)

journalAbbr: Surg J (N Y)

year: 2018

pubdate: 2018 Jan

volume: 4

issue: 1

pages: e46-e51

abstract: Late seroma formation is a rare complication after implant-based breast enlargement surgery and even less frequent after implant removal. This case report presents a case of painful recurrent seroma formation after the removal of a ruptured Poly Implants Prothèse implant. A 52-year-old patient presented herself in our clinic with a clinical history of recurrent unilateral seroma of the right breast over a period of 8 years after the initial unilateral implant removal. Removal of the remaining implant and complete bilateral capsulectomy was performed. Intraoperative findings revealed a macroscopically thickened capsule with signs of chronic inflammation on the affected side. The clinical history and the macroscopic appearance of the capsule demanded histopathological exclusion of a possible anaplastic large cell lymphoma. Histopathological and microbiological analysis of the capsule and encapsulated material revealed no signs of malignancy or infection. Immediate soft tissue reconstruction of the breast was successfully performed using autologous fat transfer. An aesthetically satisfying result regarding symmetry and volume was achieved, and no further seroma formation was observed within a 6-month follow-up period. Level of evidence: V, Case Report.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5869046/

Record 1045

pmcid: PMC6229095

pmid: 30396888

doi: 10.1136/bcr-2017-219688

title: Recurrent complications of PAAG implants during lactation.

authors

- Bourke AG
- Jose C

journal: BMJ case reports

journalAbbr: BMJ Case Rep

year: 2018

pubdate: 2018 Nov 5

volume: 2018

issue:

pages:

abstract: A 35-year-old lactating woman with pre-existing polyacrylamide gel (PAAG) implants for 10 years presented on numerous occasions following both her pregnancies with bilateral recurrent breast infection, pain and finally massive breast enlargement with a ruptured galactocoele necessitating surgical intervention. As the safety of PAAG for the breastfeeding baby is not known, breastfeeding with PAAG implants is not recommended.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6229095/

Record 1046

pmcid: PMC5869422

pmid: 29566471

doi: 10.5999/aps.2016.02047

title: Reduction mammoplasty as a treatment for symptomatic central venous stenosis.

authors

- Fok DSF
- Joethy J

journal: Archives of plastic surgery

journalAbbr: Arch Plast Surg

year: 2018

pubdate: 2018 Mar

volume: 45

issue: 2

pages: 171-176

abstract: Central venous stenosis is a rare cause of unilateral breast edema occurring in hemodialysis patients that needs to be differentiated from other differential diagnoses, including, but not limited to, inflammatory breast carcinoma, mastitis, lymphedema, and congestive heart failure. All reports of similar cases in the available literature have described improvement or resolution of the edema after treatment. Herein, we report and discuss the pathophysiology of breast edema formation in a patient who presented with massive left-sided breast edema 7 years after being diagnosed with central venous stenosis. Medical and minimally invasive therapy had not been successful, so she underwent reduction mammoplasty to relieve the symptoms.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5869422/

Record 1047

pmcid: PMC6131463

pmid: 29318957

doi: 10.1177/1557988317750989

title: Research Brief: Self-Reports of a Constellation of Persistent Antiandrogenic, Estrogenic, Physical, and Psychological Effects of Finasteride Usage Among Men.

authors

- Walf AA
- Kaurejo S
- Frye CA

journal: American journal of men's health

journalAbbr: Am J Mens Health

year: 2018

pubdate: 2018 Jul

volume: 12

issue: 4

pages: 900-906

abstract: Our research objective is to understand more, through subjective, self-reports on discussion boards/forums, persons' experiences associated with the use of drugs that alter androgen metabolism, such as finasteride. Finasteride is an orally active, specific inhibitor of 5α-reductase, which is localized to many androgen-dependent tissues. Finasteride inhibits the conversion of testosterone (T) to dihydrotestosterone (DHT), and is commonly used to treat benign prostatic hypertrophy (BPH) and male pattern baldness (MPB), both disorders associated with elevated DHT levels and 5α-reductase activity in the prostate and hair follicles, respectively. It is now acknowledged that long-term use and discontinuation of finasteride has adverse effects (AEs); however, these claims have not been well documented. In this study, discussion board posts (forums)

were analyzed as self-reports of what finasteride users indicate is problematic for them. Reports were categorized by the age of subjects as well as the types of AEs described: antiandrogenic, estrogenic, central, and nonspecific/severe. A total of 244 cases were recorded and analyzed on the discussion forum on propeciahelp.com . Among these, 74 (32%) cases reported antiandrogenic affects, 43 (19%) reported estrogenic effects, 70 (30%) reported central effects, 11 (5%) reported nonspecific/severe AEs, and 31 (14%) reported AEs in all categories. The categorization of AEs may prompt further investigation into the pathophysiology of post-finasteride syndrome (PFS). Also, subjective reports may engender greater understanding of the perceived lasting AEs of finasteride.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6131463/

Record 1048

pmcid: PMC6104423

pmid: 30167396

doi:

title: Safety Profile of Levonorgestrel: A Disproportionality Analysis of Food and Drug Administration Adverse Event Reporting System (Faers) Database.

authors

- Kurian A
- Kaushik K
- Subeesh V
- Maheswari E
- Kunnavil R

journal: Journal of reproduction & infertility

journalAbbr: J Reprod Infertil

year: 2018

pubdate: 2018 Jul-Sep

volume: 19

issue: 3

pages: 152-156

abstract: BACKGROUND: Levonorgestrel is most commonly utilized as an emergency oral contraceptive. Little is known and/or studied about the adverse effects of levonorgestrel, therefore, current investigation was aimed to generate signal for unreported adverse drug reactions of levonorgestrel using disproportionality analysis in food and drug administration adverse events reporting system database. METHODS: In FDA Adverse Events Reporting System (FAERS) database, all adverse event reports for levonorgestrel between January 2006 to June 2015 were identified and disproportionality analysis was conducted for selected adverse events of levonorgestrel using Reporting Odds Ratio, Proportional Reporting Ratio and Information Component with 95% confidence interval. RESULTS: A disproportionality analysis was done for 15 adverse events of levonorgestrel; out of these, signal for 10 adverse events was found and among them menstruation delayed was reported maximum (1791), followed by pregnancy after post-coital contraception (942), breast tenderness (901), metrorrhagia (899), dysmenorrhea (822), menorrhagia (541), nipple disorder (141), breast enlargement (77), ectopic pregnancy (61) and premenstrual syndrome (35). Pregnancy after post-coital contraception showed the highest signal having the Information Component value of 129.2, Reporting Odds Ratio value of 6.51 and Proportional Reporting Ratio value of 6.49. CONCLUSION: In this paper, ten novel AEs were identified that were disproportionately reported with the use of LNG by using data mining techniques. Although a causal relationship cannot be established, the number of cases reported suggests that there might be an association. If confirmed by epidemiologic studies, the findings from this study would have potential implications for the use of LNG and patient management in clinical practice.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6104423/

Record 1049

pmcid: PMC5888211

pmid: 29385433

doi: 10.1210/er.2017-00147

title: Sexual Dimorphism and the Origins of Human Spinal Health.

authors

- Gilsanz V
- Wren TAL
- Ponrartana S
- Mora S
- Rosen CJ

journal: Endocrine reviews

journalAbbr: Endocr Rev

year: 2018

pubdate: 2018 Apr 1

volume: 39

issue: 2

pages: 221-239

abstract: Recent observations indicate that the cross-sectional area (CSA) of vertebral bodies is on average 10% smaller in healthy newborn girls than in newborn boys, a striking difference that increases during infancy and puberty and is greatest by the time of sexual and skeletal maturity. The smaller CSA of female vertebrae is associated with greater spinal flexibility and could represent the human adaptation to fetal load in bipedal posture. Unfortunately, it also imparts a mechanical disadvantage that increases stress within the vertebrae for all physical activities. This review summarizes the potential endocrine, genetic, and environmental determinants of vertebral cross-sectional growth and current knowledge of the association between the small female vertebrae and greater risk for a broad array of spinal conditions across the lifespan.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5888211/

Record 1050

pmcid: PMC5808814

pmid: 29424285

doi: 10.1177/0306312718757081

title: Shifting syndromes: Sex chromosome variations and intersex classifications.

authors

- Griffiths DA

journal: Social studies of science

journalAbbr: Soc Stud Sci

year: 2018

pubdate: 2018 Feb

volume: 48

issue: 1

pages: 125-148

abstract: The 2006 'Consensus statement on management of intersex disorders' recommended moving to a new classification of intersex variations, framed in terms of 'disorders of sex development' or DSD. Part of the rationale for this change was to move away from associations with gender, and to increase clarity by grounding the classification system in genetics. While the medical community has largely accepted the move, some individuals from intersex activist communities have condemned it. In addition, people both inside and outside the medical community have disagreed about what should be covered by the classification system, in particular whether sex chromosome variations and the related diagnoses of Turner and Klinefelter's syndromes should be included. This article explores initial descriptions of Turner and Klinefelter's syndromes and their subsequent inclusion in intersex classifications, which were increasingly grounded in scientific understandings of sex chromosomes that emerged in the 1950s. The article questions the current drive to stabilize and 'sort out' intersex classifications through a grounding in genetics. Alternative social and historical definitions of intersex - such as those proposed by the intersex activists - have the potential to do more justice to the lived experience of those affected by such classifications and their consequences.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5808814/

Record 1051

pmcid: PMC6171257

pmid: 30327786

doi: 10.1155/2018/6784971

title: Silicone-Induced Granuloma of Breast Implant Capsule (SIGBIC): Histopathology and Radiological Correlation.

authors

- de Faria Castro Fleury E
- D'Alessandro GS
- Lordelo Wludarski SC

journal: Journal of immunology research

journalAbbr: J Immunol Res

year: 2018

pubdate: 2018

volume: 2018

issue:

pages: 6784971

abstract: Currently, attention has been given to complications related to breast implants, especially due to the presence of anaplastic large cell lymphoma (ALCL) related to silicone implants. Many manuscripts attempt to associate silicone presence with clinical complaints reported by patients, while others try to demonstrate the mechanisms of silicone bleeding by permeability loss of breast implant surfaces. There also are reports of foreign body type reactions from implant fibrous capsule to silicone corpuscles. However, there seems to be no study that correlates the clinical, radiological, and histological correlations of these lesions. The objective of this review is to correlate radiological findings of silicone-induced granuloma of breast implant capsule (SIGBIC) from breast MRI (BMRI) scans and complementary findings of ultrasound (US) and positron emission tomography (PET) scan, and its histology originated from surgical breast implant capsulectomy. To make this correlation possible, we divided SIGBIC into three radiological findings: (1) intracapsular SIGBIC, (2) SIGBIC with extracapsular extension, and (3) mixed SIGBIC associated with seroma. Our experience demonstrates histological-radiological correlation in SIGBIC diagnosis. Knowledge of these findings may demonstrate its real importance in terms of public health and patient management. We believe that SIGBIC is currently underdiagnosed by lack of training, guidance, and management in our clinical practice.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6171257/

Record 1052

pmcid: PMC6090300

pmid: 30116334

doi: 10.3892/etm.2018.6224

title: Single-incision surgery for gynecomastia using TriPort: A case report.

authors

- Liu J
- Han Y
- Cheng K
- Wang XH
- Guo F
- Yang ZL

journal: Experimental and therapeutic medicine

journalAbbr: Exp Ther Med

year: 2018

pubdate: 2018 Aug

volume: 16

issue: 2

pages: 797-801

abstract: Gynecomastia is a common benign condition of the male breast, frequently observed in newborns, adolescents and the elderly. Surgical excision remains one of the most effective methods for the management of patients with gynecomastia, particularly those with fibrosis or a course of >1 year. The present study describes two cases of patients with gynecomastia and presents a novel method of treatment, combining endoscopic surgery and liposuction using a TriPort, which may effectively remove breast tissue and shorten hospitalization time compared with a direct excision method.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6090300/

Record 1053

pmcid: PMC6316940

pmid: 30486500

doi: 10.3390/ma11122393

title: The A, B and C's of Silicone Breast Implants: Anaplastic Large Cell Lymphoma, Biofilm and Capsular Contracture.

authors

- Mempin M
- Hu H
- Chowdhury D
- Deva A
- Vickery K

journal: Materials (Basel, Switzerland)

journalAbbr: Materials (Basel)

year: 2018

pubdate: 2018 Nov 28

volume: 11

issue: 12

pages:

abstract: Breast implantation either for cosmetic or reconstructive e purposes is one of the most common procedures performed in plastic surgery. Biofilm infection is hypothesised to be involved in the development of both capsular contracture and anaplastic large cell lymphoma (ALCL). Capsular contracture is one of the principal reasons for breast revision surgery and is characterised by the tightening and hardening of the capsule surrounding the implant, and ALCL is an indolent lymphoma found only in women with textured implants. We describe the types of breast implants available with regard to their surface characteristics of surface area and roughness and how this might contribute to capsular contracture and/or biofilm formation. The pathogenesis of capsular contracture is thought to be due to biofilm formation on the implant, which results in on-going inflammation. We describe the current research into breast implant associated ALCL and how implant properties may affect its pathogenesis, with ALCL only occurring in women with textured implants.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6316940/

Record 1054

pmcid: PMC6312921

pmid: 30599483

doi: 10.6065/apem.2018.23.4.215

title: The effect of overweight on the luteinizing hormone level after gonadorelin stimulation test in girls with idiopathic central precocious puberty.

authors

- Lee HY
- Lee YJ
- Ahn MB
- Cho WK
- Suh BK

journal: Annals of pediatric endocrinology & metabolism

journalAbbr: Ann Pediatr Endocrinol Metab

year: 2018

pubdate: 2018 Dec

volume: 23

issue: 4

pages: 215-219

abstract: PURPOSE: We investigated the effect of overweight on luteinizing hormone (LH) levels after a gonadorelin stimulation test in Korean girls with idiopathic central precocious puberty (CPP). METHODS: Medical records of 234 girls diagnosed with idiopathic CPP were reviewed retrospectively. CPP was diagnosed when the peak LH levels after gonadorelin stimulation was >5.0 U/L. The enrolled girls had a peak LH level >5.0 U/L after a gonadorelin stimulation test. Selected girls were classified as normoweight (body mass index [BMI] below the 85th percentile with respect to age) and overweight (BMI greater than the 85th percentile with respect to age). RESULTS: The peak LH (8.95±2.85 U/L vs. 11.97±8.42 U/L, P<0.01) and peak follicle-stimulating hormone (9.60±2.91 U/L vs. 11.17±7.77 U/L, P=0.04) after gonadorelin stimulation were lower in overweight girls with idiopathic CPP than in normoweight girls with idiopathic CPP. Being overweight was negatively associated with peak LH levels after gonadorelin stimulation test (odds ratio, 0.89; 95 % confidence interval, 0.81-0.98, P=0.02). CONCLUSION: In girls with idiopathic CPP, being overweight led to a lower LH peak after

gonadorelin stimulation. Further research is needed to better understand the role of overweight on gonadotropin secretion in precocious puberty.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6312921/

Record 1055

pmcid: PMC6279436

pmid: 30509934

doi: 10.1042/EBC20170053

title: The genetic basis of disease.

authors

- Jackson M
- Marks L
- May GHW
- Wilson JB

journal: Essays in biochemistry

journalAbbr: Essays Biochem

year: 2018

pubdate: 2018 Dec 3

volume: 62

issue: 5

pages: 643-723

abstract: Genetics plays a role, to a greater or lesser extent, in all diseases. Variations in our DNA and differences in how that DNA functions (alone or in combinations), alongside the environment (which encompasses lifestyle), contribute to disease processes. This review explores the genetic basis of human disease, including single gene disorders, chromosomal imbalances, epigenetics, cancer and complex disorders, and considers how our understanding and technological advances can be applied to provision of appropriate diagnosis, management and therapy for patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6279436/

Record 1056

pmcid: PMC5865941

pmid: 29616164

doi: 10.1097/GOX.0000000000001649

title: The Impact of Recipient Site External Expansion in Fat Grafting Surgical Outcomes.

authors

- Oranges CM
- Striebel J
- Tremp M
- Madduri S
- Kalbermatten DF
- Schaefer DJ

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2018

pubdate: 2018 Feb

volume: 6

issue: 2

pages: e1649

abstract: BACKGROUND: The fat grafting process includes the 4 phases of tissue harvesting, processing, recipient-site preparation, and reinjection. Among them, the preparation of the recipient site has never been exhaustively reviewed. We aim to provide a comprehensive overview of the methods to prepare the recipient site through external expansion with the resulting outcomes. METHODS: PubMed/Medline database was searched for studies on fat grafting recipient site preparation by applying the following algorithm: ((fat grafting) OR (lipofilling) OR (lipograft) AND (recipient site)). A priori criteria were used to review the resulting articles and identify those dealing with external expansion. RESULTS: Fourteen studies published from 2008 through 2016 met inclusion criteria (4 case reports, 6 retrospective, and 4 prospective studies), representing 1,274 treated patients. Two devices for preexpansion were used with different protocols: BRAVA system and Kiwi VAC-6000M with a PalmPump. The 13 studies that applied the BRAVA system reported large fat volume transplantation to the breast (average > 200 cc). The most common complications were localized edema (14.2%), temporary bruising, and superficial skin blisters (11.3%), while the most serious was pneumothorax (0.5%). The majority of the studies reported enhancement of fat graft survival, which ranged between 53% and 82% at 6 months to 1 year follow-up, and high satisfaction of patients and surgeon. CONCLUSIONS: External expansion and fat grafting is a promising technique for breast reconstruction and augmentation. However, due to the overall low level of evidence of the available studies, further research is needed to validate the procedure.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5865941/

Record 1057

pmcid: PMC5780290

pmid: 29379733

doi: 10.1016/j.ajur.2017.11.005

title: The use of 5-alpha reductase inhibitors in the treatment of benign prostatic hyperplasia.

authors

- Kim EH
- Brockman JA
- Andriole GL

journal: Asian journal of urology

journalAbbr: Asian J Urol

year: 2018

pubdate: 2018 Jan

volume: 5

issue: 1

pages: 28-32

abstract: Benign prostatic hyperplasia (BPH) is characterized by an enlarged prostate, lower urinary tract symptoms (LUTS), and a decreased urinary flow rate. Common in older men, BPH is a progressive disease that can eventually lead to complications including acute urinary retention (AUR) and the need for BPH-related surgery. Both normal and abnormal prostate growth is driven by the androgen dihydrotestosterone (DHT), which is formed from testosterone under the influence of 5-alpha reductase. Thus, 5-alpha reductase inhibitors (5-ARIs) effectively reduce the serum and intraprostatic concentration of DHT, causing an involution of prostate tissue. Two 5-ARIs are currently available for the treatment of BPH-finasteride and dutasteride. Both have been

demonstrated to decrease prostate volume, improve LUTS and urinary flow rates, which ultimately reduces the risk of AUR and BPH-related surgery. Therefore, either alone or in combination with other BPH medications, 5-ARIs are a mainstay of BPH management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5780290/

Record 1058

pmcid: PMC5838282

pmid: 29531723

doi: 10.1002/ccr3.1362

title: Virginal breast hypertrophy in a patient with Beckwith-Wiedemann syndrome.

authors

- Szymańska E
- Moszczyńska E
- Polnik D
- Szymańska S
- Jurkiewicz E
- Pyzlak M
- Armata M
- Walewska-Wolf M
- Kaliciński P
- Rokicki D
- Szalecki M

journal: Clinical case reports

journalAbbr: Clin Case Rep

year: 2018

pubdate: 2018 Mar

volume: 6

issue: 3

pages: 484-489

abstract: Virginal breast hypertrophy is a multidisciplinary condition including surgical, pediatric, and endocrine/gynecological disciplines, and its successful diagnosis and management requires complex, team approach.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5838282/

Record 1059

pmcid: PMC5471982

pmid:

doi: 10.1017/mdh.2017.37

title: .

authors

- Bourke J

journal: Medical History

journalAbbr: Med Hist

year: 2017

pubdate: 2017 Jul

volume: 61

issue: 3

pages: 444-5

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5471982/

Record 1060

pmcid: PMC5554642

pmid: 28819518

doi: 10.11604/pamj.2017.27.97.12484

title: [A rare case of giant galactocele associated with prolactinoma].

authors

- Tayae M
- Jamor J

journal: The Pan African medical journal

journalAbbr: Pan Afr Med J

year: 2017

pubdate: 2017

volume: 27

issue:

pages: 97

abstract: Galactocele is a rare cystic formation, a benign breast lesion, occurring when breast duct is blocked and engorged. It generally affects postpartum women, either breastfeeding or not. Only a few cases have been reported in the literature and they were not related to lactation, as in the case of postmenopausal women or of men; moreover, their relationship to the overproduction of prolactin, a growth factor stimulating mammary epithelial cells, is not very well defined at this time. We here report the unusual case of a 30-year old patient with no personal history of childbirth or abortion. She was treated in the Division of Endocrinology for pituitary microadenoma with Cabergoline, that she stopped for 1 year. Even taking into account this rare association, it is important to emphasize the role of hormones in the progression of breast anatomy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5554642/

Record 1061

pmcid: PMC5769836

pmid: 29301188

doi: 10.6065/apem.2017.22.4.266

title: A boy with 46,X,+mar presenting gynecomastia and short stature.

authors

- Kim KE
- Kim YJ
- Jung MK

- Chae HW
- Kwon AR
- Lee WJ
- Kim DH
- Kim HS

journal: Annals of pediatric endocrinology & metabolism

journalAbbr: Ann Pediatr Endocrinol Metab

year: 2017

pubdate: 2017 Dec

volume: 22

issue: 4

pages: 266-271

abstract: A 15-year-old boy was referred due to gynecomastia and short stature. He was overweight and showed the knuckle-dimple sign on the left hand, a short fourth toe on the left foot, and male external genitalia with a small phallus. His levels of estradiol and follicle-stimulating hormone were increased, and his testosterone concentration was normal. Other hormonal tests were within the normal range. Radiographs showed short fourth and fifth metacarpals and fourth metatarsal bones. The karyotype was reported as 46,X,+mar, and the marker chromosome was shown to originate from the Y chromosome, which was identified by fluorescence in situ hybridization. Polymerase chain reaction and direct sequencing were used to clarify the deleted loci of the Y chromosome by making use of Y-specific sequence-tagged sites (STSs). The sex-determining region Y and centromere were verified, and there were microdeletions on the long arm of the Y chromosome. The azoospermia factor (AZF) b region was partially deleted, and AZFa and AZFc were completely deleted. Two STS probes of sY143 and the Y chromosome RNA recognition motif in AZFb showed positive signals corresponding to Yq11.223. The karyotype of the patient was interpreted as 46,X,der(Y)del(Y)(q11.21q11.222)del(Y)(q11.23qter). Herein, we report a rare case of a boy presenting with gynecomastia and short stature with 46, X, +mar, which originated from the Y chromosome, which was identified to have Yq microdeletions.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5769836/

Record 1062

pmcid: PMC5579424

pmid: 28904695

doi: 10.11604/pamj.2017.27.167.11281

title: A case of gestational gigantomastia in a 37-years-old woman associated with elevated ANA: a casual linkage?

authors

- Zingaretti N
- De Biasio F
- Riccio M
- Nardini N
- Mariuzzi L
- Parodi PC

journal: The Pan African medical journal

journalAbbr: Pan Afr Med J

year: 2017

pubdate: 2017

volume: 27

issue:

pages: 167

abstract: Hypertrophy of the breast (macromastia and gigantomastia) is a rare medical condition of the breast connective tissues. The etiology of this condition is still not clear; rarely, gigantomastia has been reported to develop in the setting of an autoimmune illness. We reported a case of a 37-years-old woman with undifferentiated connective tissue disease of 2-years duration presented with enlargement of breasts. The breast enlargment started at 5 months of gestation. She successfully underwent reduction mammoplasty with free nipple graft. In the succeeding months the level of antinuclear ANA remained stable. It is uncertain whether a positive antinuclear antibodies in gigantomastia is a casuative agent or an effect.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5579424/

Record 1063

pmcid: PMC5799937

pmid: 29430160

doi: 10.4103/jhrs.JHRS_128_17

title: A Case of Primary Amenorrhea with Swyer Syndrome.

authors

- Priya PK
- Mishra VV
- Choudhary S
- Rizvi JS

journal: Journal of human reproductive sciences

journalAbbr: J Hum Reprod Sci

year: 2017

pubdate: 2017 Oct-Dec

volume: 10

issue: 4

pages: 310-312

abstract: Swyer syndrome with complete gonadal dysgenesis is associated with an absence of testicular differentiation in a phenotypic female with a 46, XY karyotype. A 14-year-old unmarried girl was referred with complaints of primary amenorrhea and nondevelopment of breast. Her built was normal. Examination of her secondary sexual characteristics revealed no breast development, absent axillary hairs, and sparse pubic hairs. External genitalia was of female type. Karyotype showed genotype of 46, XY. Magnetic resonance imaging revealed hypoplastic uterus with absent fallopian tubes and ovaries. A diagnosis of Swyer syndrome was made. Laparoscopy showed infantile uterus, normal fallopian tubes, and streak gonads. Laparoscopic removal of streak gonads was done as there is a risk of gonadoblastoma in such cases. The patient was started on hormonal replacement therapy. Swyer syndrome results mainly due to mutation in certain genes such as SRY gene, which leads to failure of development of testis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5799937/

Record 1064

pmcid: PMC5702868

pmid: 29546008

doi: 10.1016/j.ijscr.2017.11.026

title: A prepubertal giant juvenile fibroadenoma in a 12-year-old girl: Case report and brief literature review.

authors

- Giannos A
- Stavrou S
- Gkali C
- Chra E
- Marinopoulos S
- Chalazonitis A
- Dimitrakakis C
- Drakakis P

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2017

pubdate: 2017

volume: 41

issue:

pages: 427-430

abstract: INTRODUCTION: Giant juvenile fibroadenomas represent only the 0.5% of all fibroadenomas, constituting a rare condition in adolescence. In prepuberty, the presence of this condition is extremely rare. PRESENTATION OF CASE: We describe a rare case of a 12- year-old Caucasian girl who presented to our Hospital complaining of a palpable mass with rapid enlargement in her right breast that she had first noticed 3 months ago. Her menarche hadn't occurred yet. DISCUSSION: Physical examination showed a giant mass of 15 × 13 cm in the right breast. The patient was further evaluated via ultrasonography showing a sole large lesion of 13 × 12 cm in the right breast. A surgical procedure under general anesthesia was performed. Histopathological findings after the surgical excision were suggestive of giant juvenile fibroadenoma. The patient has a normal breast development over a period of 9 month follow up. CONCLUSION: Giant juvenile fibroadenomas should be included in differential diagnosis of a breast mass in prepubertal girls despite the fact that they are very rare in prepuberty, tend to appear later during adolescence and their prevalence is lower in Caucasians. The remarkable size and the rapid growth of the lesion should not be ruled out in the diagnostic process of an adolescent breast.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5702868/

Record 1065

pmcid: PMC6754040

pmid:

doi: 10.1515/iss-2017-2007

title: Abstracts DGPRAEC.

authors

journal: Innovative Surgical Sciences

journalAbbr: Innov Surg Sci

year: 2017

pubdate: 2017 Mar

volume: 2

issue: Suppl 1

pages: s291-309

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6754040/

Record 1066

pmcid: PMC5240059

pmid: 28217517

doi: 10.4103/2230-8210.196022

title: Adolescent gynecomastia is associated with a high incidence of obesity, dysglycemia, and family background of diabetes mellitus.

authors

- Kulshreshtha B
- Arpita A
- Rajesh PT
- Sameek B
- Dutta D
- Neera S
- Mohd M

journal: Indian journal of endocrinology and metabolism

journalAbbr: Indian J Endocrinol Metab

year: 2017

pubdate: 2017 Jan-Feb

volume: 21

issue: 1

pages: 160-164

abstract: BACKGROUND: Gynecomastia during adolescence is common though etiology is not clear. We studied the clinical and hormonal profile of adolescent patients with gynecomastia. METHODOLOGY: Patients who had onset of breast development between age 10 and 20 years were included in this study. Their clinical profile, biochemical, and hormonal parameters were studied. RESULTS: Of 94 patients with gynecomastia, 4 had hypogonadotropic hypogonadism, 4 had hypergonadotropic hypogonadism, and 1 had fibroadenosis, but in majority (90.4%), no apparent cause for breast enlargement was evident. In the idiopathic group, majority were obese (63%). Fourteen (16%) patients had impaired fasting glucose or impaired glucose tolerance. Another twenty patients had subtle abnormalities (high 1 h glucose or glucose peak at 2 h). Twenty-nine percent of lean and 38% of obese patients had mild abnormalities in glucose profile. Sixty percent of patients had family background of diabetes. Obese patients had lower testosterone as compared to lean patients; however, estradiol, luteinizing hormone, and follicle-stimulating hormone levels were similar in the two groups. CONCLUSION: Gynecomastia during adolescence is associated with obesity, dysglycemia, and family background of diabetes mellitus.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5240059/

Record 1067

pmcid: PMC5843236

pmid: 29568644

doi: 10.4102/sajhivmed.v18i1.776

title: Adult antiretroviral therapy guidelines 2017.

authors

- Meintjes G
- Moorhouse MA
- Carmona S
- Davies N
- Dlamini S
- van Vuuren C
- Manzini T
- Mathe M
- Moosa Y
- Nash J
- Nel J
- Pakade Y
- Woods J
- Van Zyl G
- Conradie F
- Venter F

journal: Southern African journal of HIV medicine

journalAbbr: South Afr J HIV Med

year: 2017

pubdate: 2017

volume: 18

issue: 1

pages: 776

abstract: These guidelines are intended as an update to those published in the Southern African Journal of HIV Medicine in 2014 and the update on when to initiate antiretroviral therapy in 2015. Since the release of the previous guidelines, the scale-up of antiretroviral therapy (ART) in southern Africa has continued. New antiretroviral drugs have become available with improved efficacy, safety and robustness. The guidelines are intended for countries in the southern African region, which vary between lower and middle income.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5843236/

Record 1068

pmcid: PMC5733180

pmid: 29333311

doi: 10.1155/2017/7603603

title: An Uncommon Case of Bilateral Breast Enlargement Diagnosed as Tumoral Pseudoangiomatous Stromal Hyperplasia: Imaging and Pathological Findings.

authors

- Sollozo-Dupont I
- Domínguez-Hernández HA

- Pavón-Hernández C
- Villaseñor-Navarro Y
- Shaw-Dulin R
- Pérez-Sánchez VM
- España-Ferrufino AJ
- Álvarez-Guadarrama LM
- Porras-Reyes FI
- Pérez-Badillo MP

journal: Case reports in radiology

journalAbbr: Case Rep Radiol

year: 2017

pubdate: 2017

volume: 2017

issue:

pages: 7603603

abstract: The incidence of reported pseudoangiomatous stromal hyperplasia (PASH), as well as the variability and severity of clinical presentations, is increasing in the literature. In parallel, several authors posit the need for an improved classification of PASH to avoid possible variables associated with this diagnosis. Here, we present a 25-year-old woman with PASH accompanied by severe bilateral and symmetrical breasts enlargement, highlighting an uncommon clinical presentation of PASH as much as the careful interdisciplinary review and correlation of histology and all available imaging studies to confirm the definitive diagnosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5733180/

Record 1069

pmcid: PMC5696988

pmid: 28660825

doi: 10.1308/rcsann.2017.0100

title: An unexpected diagnosis of breast malignancy.

authors

- Richardson T
- Cottier F

journal: Annals of the Royal College of Surgeons of England

journalAbbr: Ann R Coll Surg Engl

year: 2017

pubdate: 2017 Jul

volume: 99

issue: 6

pages: e180-e182

abstract: Inflammatory breast cancer constitutes 5% of all breast cancer diagnoses. Diagnosis is based on clinical signs including skin changes, erythema and oedema, together with rapid progression and involvement of more than one-third of the affected breast. It is an aggressive tumour with great metastatic potential, metastases being present in 30% of patients at first presentation. Primary non-Hodgkin's lymphoma of the breast is rare but is well reported. It accounts for 0.5% of all breast malignancies and 1% of all non-Hodgkin's diagnoses.

Prognosis of primary breast lymphoma varies depending on the stage of disease with stage IE having a 5-year survival rate of 78-83% and stage IIE having a 5-year survival rate of 20-57%. We present a rare case of non-Hodgkin's lymphoma mimicking an inflammatory breast cancer. The aim of this case report is to highlight an unusual presentation of non-Hodgkin's lymphoma and the diagnostic difficulties that arise.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5696988/

Record 1070

pmcid: PMC5537216

pmid: 28804211

doi: 10.1297/cpe.26.189

title: Apparent cyclic vaginal bleeding in a child: factitious disorder.

authors

- Ucakturk A
- Gunindi F
- Aydin M

journal: Clinical pediatric endocrinology : case reports and clinical investigations : official journal of the Japanese Society for Pediatric Endocrinology

journalAbbr: Clin Pediatr Endocrinol

year: 2017

pubdate: 2017

volume: 26

issue: 3

pages: 189-192

abstract: A 20-mo-old girl was brought to our department by her mother because of breast enlargement. She was diagnosed with premature thelarche. One month later, she returned to our hospital with a complaint of vaginal bleeding. During the subsequent 6 mo, her vaginal bleeding recurred every month while her breast development disappeared. We performed laboratory tests and imaging. At the end of 6 mo, we realized that her mother's menstrual bleeding and the patient's blood staining were concurrent. The mother confessed applying her vaginal flow to her daughter's underwear. Factitious disorder should be included in the differential diagnosis of unexplained vaginal bleeding in childhood.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5537216/

Record 1071

pmcid: PMC5669539

pmid: 29033391

doi: 10.5152/dir.2017.17033

title: Breast diseases in children: the spectrum of radiologic findings in a cohort study.

authors

- Durmaz E
- Öztek MA
- Arıöz Habibi H
- Kesimal U
- Sindel HT

journal: Diagnostic and interventional radiology (Ankara, Turkey)

journalAbbr: Diagn Interv Radiol

year: 2017

pubdate: 2017 Nov-Dec

volume: 23

issue: 6

pages: 407-413

abstract: PURPOSE: We aimed to investigate the spectrum of radiologic findings and referral reasons for breast diseases in children considering age-appropriate presentation. METHODS: Our retrospective cohort study included 348 consecutive pediatric patients aged <19 years (median, 13 years) referred to radiology with a clinical presentation between 2005 and 2016. Radiologic findings were reviewed in four age ranges (0-2 years, 2-8 years, 8-15 years, >15 years). RESULTS: Of 348 patients, 257 had a referral reason. The most frequent referral reason was a palpable mass (35%). Developmental abnormalities accounted for 48% of all radiologic findings in 348 patients. We did not detect any breast malignancy. According to age groups, the most common radiologic findings were neonatal hypertrophy (0-2 years), early breast development (2-8 years), developmental abnormalities by a majority of gynecomastia (8-15 years), and normal findings or developmental abnormalities (>15 years). Interestingly, the frequency of gynecomastia was only 4% in neonatal period or early childhood. Fibroadenomas and fibroadenoma-like solid masses were seen after 8 years and constituted the majority of solid masses (65%). Cysts were seen at a rate of 7% and majority of them were of simple type, which tends to resolve in time. CONCLUSION: In our study, the most common referral reason to radiology was a palpable breast mass. Neonatal hypertrophy and early breast development in younger children, and developmental abnormalities in older children may be kept in mind as the most common radiologic findings. Our study confirms the substantial absence of malignancies in children as well as a widely different disease spectrum in comparison with the adult population.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5669539/

Record 1072

pmcid: PMC5460956

pmid: 28559535

doi: 10.12659/ajcr.903161

title: Breast Implant-Associated Anaplastic Large Cell Lymphoma (ALCL): A Case Report.

authors

- Evren S
- Khoury T
- Neppalli V
- Cappuccino H
- Hernandez-Ilizaliturri FJ
- Kumar P

journal: The American journal of case reports

journalAbbr: Am J Case Rep

year: 2017

pubdate: 2017 May 31

volume: 18

issue:

pages: 605-610

abstract: BACKGROUND Anaplastic large cell lymphomas (ALCL) are a rare type of primary breast lymphoma.

The association between breast implants and ALCL was first described in 1997 and since then 34-173 cases have been presented. The annual incidence of breast implant-associated ALCL (BI-ALCL) is 0.1-0.3 per 100 000 women who undergo breast reconstruction, and cases are often underreported due to the rarity of these tumors. BI-ALCL arises from the inflammatory T cells surrounding the fibrous capsule, and most tumors are in situ. CASE REPORT Here, we present the case of a 51-year-old woman with ALCL following bilateral silicone breast implants. The patient presented with breast enlargement and tenderness 9 years following reconstructive surgery. Imagining studies showed fluid collection surrounding the affected breast implant. Staging studies and histocytopathology examination confirmed the presence BI-ALCL without capsular invasion or metastasis. Complete surgical excision was performed. The patient continues to be in complete remission. CONCLUSIONS Due to the rarity of these tumors, establishing the diagnosis of BI-ALCL can be challenging and requires a multidisciplinary approach. Clinicians should be aware of the relationship between breast implants and BI-ALCL.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5460956/

Record 1073

pmcid: PMC5457777

pmid: 28607534

doi: 10.1155/2017/4754827

title: Challenging a Misnomer? The Role of Inflammatory Pathways in Inflammatory Breast Cancer.

authors

- Morrow RJ
- Etemadi N
- Yeo B
- Ernst M

journal: Mediators of inflammation

journalAbbr: Mediators Inflamm

year: 2017

pubdate: 2017

volume: 2017

issue:

pages: 4754827

abstract: Inflammatory breast cancer is a rare, yet highly aggressive form of breast cancer, which accounts for less than 5% of all locally advanced presentations. The clinical presentation of inflammatory breast cancer often differs significantly from that of noninflammatory breast cancer; however, immunohistochemistry reveals few, if any, distinguishing features. The more aggressive triple-negative and HER2-positive breast cancer subtypes are overrepresented in inflammatory breast cancer compared with noninflammatory breast cancer, with a poorer prognosis in response to conventional therapies. Despite its name, there remains some controversy regarding the role of inflammation in inflammatory breast cancer. This review summarises the current molecular evidence suggesting that inflammatory signaling pathways are upregulated in this disease, including NF- κ B activation and excessive IL-6 production among others, which may provide an avenue for novel therapeutics. The role of the tumor microenvironment, through tumor-associated macrophages, infiltrating lymphocytes, and cancer stem cells is also discussed, suggesting that these tumor extrinsic factors may help account for the differences in behavior between inflammatory breast cancer and noninflammatory breast cancer. While there are various novel treatment strategies already underway in clinical trials, the need for further development of preclinical models of this rare but aggressive disease is paramount.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5457777/

Record 1074

pmcid: PMC5770907

pmid: 29284718

doi: 10.1136/bmjopen-2017-018121

title: Cohort profile: Study of Transition, Outcomes and Gender (STRONG) to assess health status of transgender people.

authors

- Quinn VP
- Nash R
- Hunkeler E
- Contreras R
- Cromwell L
- Becerra-Culqui TA
- Getahun D
- Giammattei S
- Lash TL
- Millman A
- Robinson B
- Roblin D
- Silverberg MJ
- Slovis J
- Tangpricha V
- Tolsma D
- Valentine C
- Ward K
- Winter S
- Goodman M

journal: BMJ open

journalAbbr: BMJ Open

year: 2017

pubdate: 2017 Dec 27

volume: 7

issue: 12

pages: e018121

abstract: PURPOSE: The Study of Transition, Outcomes and Gender (STRONG) was initiated to assess the health status of transgender people in general and following gender-affirming treatments at Kaiser Permanente health plans in Georgia, Northern California and Southern California. The objectives of this communication are to describe methods of cohort ascertainment and data collection and to characterise the study population. PARTICIPANTS: A stepwise methodology involving computerised searches of electronic medical records and free-text validation of eligibility and gender identity was used to identify a cohort of 6456 members with first evidence of transgender status (index date) between 2006 and 2014. The cohort included 3475 (54%) transfeminine (TF), 2892 (45%) transmasculine (TM) and 89 (1%) members whose natal sex and gender identity remained undetermined from the records. The cohort was matched to 127 608 enrollees with no transgender evidence (63 825 women and 63 783 men) on year of birth, race/ethnicity, study site and membership year of the index date. Cohort follow-up extends through the end of 2016. FINDINGS TO DATE: About 58% of TF and 52% of TM cohort members received hormonal therapy at Kaiser Permanente. Chest surgery was more

common among TM participants (12% vs 0.3%). The proportions of transgender participants who underwent genital reconstruction surgeries were similar (4%-5%) in the two transgender groups. Results indicate that there are sufficient numbers of events in the TF and TM cohorts to further examine mental health status, cardiovascular events, diabetes, HIV and most common cancers. FUTURE PLANS: STRONG is well positioned to fill existing knowledge gaps through comparisons of transgender and reference populations and through analyses of health status before and after gender affirmation treatment. Analyses will include incidence of cardiovascular disease, mental health, HIV and diabetes, as well as changes in laboratory-based endpoints (eg, polycythemia and bone density), overall and in relation to gender affirmation therapy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5770907/

Record 1075

pmcid: PMC5700481

pmid: 29250442

doi: 10.1155/2017/7356362

title: Conceptualising the Factors Affecting Retention in Care of Patients on Antiretroviral Treatment in Kabwe District, Zambia, Using the Ecological Framework.

authors

- Mukumbang FC
- Mwale JC
- van Wyk B

journal: AIDS research and treatment

journalAbbr: AIDS Res Treat

year: 2017

pubdate: 2017

volume: 2017

issue:

pages: 7356362

abstract: BACKGROUND: HIV remains a major public health challenge in Zambia. The roll-out of antiretroviral treatment (ART) has engendered new challenges in retention in care. OBJECTIVE: To conceptualise the factors affecting retention in care of ART patients at three primary healthcare facilities using the ecological framework. METHOD: Qualitative data were collected through in-depth interviews with 45 ART patients and three focus group discussions with 20 healthcare providers from three primary healthcare facilities in Kabwe district, Zambia, and subjected to thematic content analysis. RESULTS: Individual level barriers to retention in care included side effects, gaining weight, belief in faith healing, and use of herbal remedies and alcohol. Interpersonal barriers such as stigma and nondisclosure of HIV status were reported. At the institutional level, inadequate space in the clinic, long waiting times, long travel distances, and shortage of third-line drugs presented barriers to retention in care. Food shortages and patient mobility were reported as community barriers to retention in care. CONCLUSION: The ecological framework conceptualises the complex and dynamic factors affecting retention in ART care and highlights the need for multifaceted interventions that combine health education, disease management, and opportunities for income generation in a socially responsive and accountable environment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5700481/

Record 1076

pmcid: PMC5419004

pmid: 28529378

doi: 10.4103/psychiatry.IndianJPsychiatry_307_16

title: Delusion of pregnancy in a 70-year-old male.

authors

- Gaikwad BH
- Dharmadhikari AR
- Subramanyam AA
- Kedare JS
- Kamath RM

journal: Indian journal of psychiatry

journalAbbr: Indian J Psychiatry

year: 2017

pubdate: 2017 Jan-Mar

volume: 59

issue: 1

pages: 130

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5419004/

Record 1077

pmcid: PMC5750303

pmid: 29302481

doi: 10.21037/gs.2017.08.07

title: Development of a surgical algorithm by using pre-operative imaging to predict mammoplasty cosmetic outcomes for large non-malignant tumours.

authors

- Lim GH
- Ng RP
- Leong LCH

journal: Gland surgery

journalAbbr: Gland Surg

year: 2017

pubdate: 2017 Dec

volume: 6

issue: 6

pages: 649-653

abstract: BACKGROUND: Use of mammoplasty for resection of large non-malignant tumours is not widely described. We aim to determine the optimal tumour to breast size ratio cut-off in this group of patients undergoing mammoplasty which could achieve satisfactory cosmetic outcomes. METHODS: Patients who underwent mammoplasty from May 2014 to June 2017 for biopsy-proven large non-malignant tumours were included in the study. The demographics, tumour to breast size ratios, histological features and cosmetic outcomes of these patients were assessed. The tumour to breast size ratio was estimated in ten-percentiles of the relative volume of the tumour to the overall breast size based on ultrasound and/or mammogram images. RESULTS: Ten patients were recruited. Median age was 40 years old (range, 14-55 years old). One, four and five patients underwent vertical, wise pattern and round block mammoplasties respectively. The median tumour

to breast size ratio was 55% (range, 20-90%). Histology revealed phyllodes and giant fibroadenoma in six and four patients respectively. Margins were clear in all cases. Mean weight and average maximum size of surgical specimen was 357.9 g (range, 28-1,186 g) and 99.3 mm (range, 35-165 mm) respectively. One patient developed partial nipple necrosis which was treated conservatively. All the patients with an estimated tumour to breast size ratio up to 70% reported good to excellent cosmetic outcomes, except for one patient who had a large tumour occupying 90% of her breast. CONCLUSIONS: Mammoplasty can be used successfully in patients with large non-malignant tumours. However, in patients with tumours occupying more than 70% of the breast, mammoplasty alone may not yield a good cosmetic outcome.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5750303/

Record 1078

pmcid: PMC5560510

pmid: 28839450

doi: 10.1177/1179545X17725034

title: Diffuse Large B-Cell Breast Lymphoma: A Case Series.

authors

- Al Battah AH
- Al Kuwari EA
- Hascsi Z
- Nashwan AJ
- Elomari H
- Elsabah H
- Al Azawi S
- Kohla S
- Soliman D
- Yassin MA

journal: Clinical medicine insights. Blood disorders

journalAbbr: Clin Med Insights Blood Disord

year: 2017

pubdate: 2017

volume: 10

issue:

pages: 1179545X17725034

abstract: Primary breast lymphoma (PBL) is a rare disease, and few clinicohistopathologic features of the disease have been discussed in previous studies. It represents 2.2% of extranodal lymphomas and constitutes 0.04% to 0.5% of malignant breast neoplasms, despite the clinical and radiographic similarities between breast lymphoma and carcinoma, the prognosis, as reported in the literature, varies. No consensus exists on the best way to treat PBL. However, radiotherapy and chemotherapy were used alone or in combination to treat various cases of PBL. We retrospectively studied 3 cases of PBL of the breast seen in patients attending a tertiary cancer center in Qatar, between 2012 and 2015, in an attempt to determine the common clinical features, therapy, and prognosis of PBL.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5560510/

Record 1079

pmcid: PMC5767612

pmid: 29114697

doi: 10.15537/smj.2017.11.21187

title: Effect of gonadotropin-releasing hormone analogue on final adult height among Jordanian children with precocious puberty.

authors

- Swaiss HH
- Khawaja NM
- Farahid OH
- Batieha AM
- Ajlouni KM

journal: Saudi medical journal

journalAbbr: Saudi Med J

year: 2017

pubdate: 2017 Nov

volume: 38

issue: 11

pages: 1101-1107

abstract: To study the effect of gonadotropin-releasing hormone analogue (GnRHa) on final adult height (FAH) among Jordanian children with central precocious puberty (CPP). Methods: It is a retrospective historical cohort study. We assessed the FAH and height gain in 43 children with CPP (39 females and 4 males) who received GnRHa and 13 children with CPP (11 females and 2 males) who did not receive GnRHa and achieved FAH between 2004 and 2014. Final adult height was compared to target height (TH) and mid- parental height (MPH) in both groups. Results: In GnRHa treated females, the FAH was 158.5±6.6 cm compared to 151.2±8.4 cm in the untreated females (p=0.004). Height gain was 2.9±8.5 cm in the treated females compared to -3.8±7.7 cm in the untreated group (p=0.022). In GnRHa treated females, FAH was found to be closer to TH (p=0.01) and MPH (p=0.01) in comparison to untreated females. Conclusion: Gonadotropin-releasing hormone analogue is effective in increasing FAH in Jordanian children with CPP, particularly those with advanced bone age.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5767612/

Record 1080

pmcid: PMC5580378

pmid: 28319558

doi: 10.1097/PSY.0000000000000465

title: Effects of Medical Interventions on Gender Dysphoria and Body Image: A Follow-Up Study.

authors

- van de Grift TC
- Elaut E
- Cerwenka SC
- Cohen-Kettenis PT
- De Cuypere G
- Richter-Appelt H
- Kreukels BPC

journal: Psychosomatic medicine

journalAbbr: Psychosom Med

year: 2017

pubdate: 2017 Sep

volume: 79

issue: 7

pages: 815-823

abstract: OBJECTIVE: The aim of this study from the European Network for the Investigation of Gender Incongruence is to investigate the status of all individuals who had applied for gender confirming interventions from 2007 to 2009, irrespective of whether they received treatment. The current article describes the study protocol, the effect of medical treatment on gender dysphoria and body image, and the predictive value of (pre)treatment factors on posttreatment outcomes. METHODS: Data were collected on medical interventions, transition status, gender dysphoria (Utrecht Gender Dysphoria Scale), and body image (Body Image Scale for transsexuals). In total, 201 people participated in the study (37% of the original cohort). RESULTS: At follow-up, 29 participants (14%) did not receive medical interventions, 36 hormones only (18%), and 136 hormones and surgery (68%). Most transwomen had undergone genital surgery, and most transmen chest surgery. Overall, the levels of gender dysphoria and body dissatisfaction were significantly lower at follow-up compared with clinical entry. Satisfaction with therapy responsive and unresponsive body characteristics both improved. High dissatisfaction at admission and lower psychological functioning at follow-up were associated with persistent body dissatisfaction. CONCLUSIONS: Hormone-based interventions and surgery were followed by improvements in body satisfaction. The level of psychological symptoms and the degree of body satisfaction at baseline were significantly associated with body satisfaction at follow-up.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5580378/

Record 1081

pmcid: PMC5617685

pmid: 28930818

doi: 10.1097/MD.0000000000007619

title: Factors affecting breastfeeding adherence among Chinese mothers: A multicenter study.

authors

- Huang P
- Ren J
- Liu Y
- Luo B
- Zhao X

journal: Medicine

journalAbbr: Medicine (Baltimore)

year: 2017

pubdate: 2017 Sep

volume: 96

issue: 38

pages: e7619

abstract: Breastfeeding is beneficial for both infant and mother, but discontinuation of breastfeeding is very common.To investigate maternal breastfeeding intention and the rate of breastfeeding based on the theory of reasoned action, and analyze the predominant factors associated with breastfeeding and breastfeeding problems.This observational study was conducted in 3 hospitals. Three researchers recruited women at 3 time

points in the hospitals: initial documentation of pregnancy at the outpatient department, prenatal admission, and postpartum discharge. SPSS version 21 was used for statistical analyses. Significance was set at P < .05. In the multivariate analysis, binary logistic regression was used and odds ratios (ORs) with 95% confidence intervals (CI) were calculated.We recruited 1260 women, with 420 pregnant women at each time point. 55.1% of the infants were exclusively breastfed, 40.6% were mixed fed, and 4.3% were formula fed when discharged from hospital. A total of 53.8% of the mothers declared having breastfeeding problems. The multivariate analysis showed that nonsuccessful breastfeeding was associated with neonatal birth length, food intake before breastfeeding, infrequent sucking, the intention of breastfeeding, understanding level of the benefits of breastfeeding and that breastfeeding problems were related with the understanding level of the benefits of breastfeeding, neonatal birth length, normal vaginal delivery, breast size, the experience of breastfeeding, use of pacifier and the needs of family member's support in breastfeeding.Most mothers who intended to practice exclusive breastfeeding initially chose to add formula and had breastfeeding problems when discharged from hospital. Successful breastfeeding depends on antenatal and postnatal breastfeeding education and on support provided by healthcare professionals.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5617685/

Record 1082

pmcid: PMC5534236

pmid: 28804590

doi: 10.5001/omj.2017.66

title: Feminizing Adrenocortical Carcinoma Without Gynecomastia.

authors

- Chentli F
- Chabour F
- Bouchibane D
- Nouar N

journal: Oman medical journal

journalAbbr: Oman Med J

year: 2017

pubdate: 2017 Jul

volume: 32

issue: 4

pages: 349-351

abstract: Malignant feminizing adrenocortical tumors are exceedingly rare. Their main presentation is gynecomastia. In these estrogen secreting tumors (with or without other adrenocortical hormones) lack of gynecomastia is exceptional as in our case. A 44-year-old man presented with abdominal pain. Radiological assessment revealed a tumor measuring 120 × 95 mm in the retroperitoneal area with numerous metastases. Pathological examination pleaded for an adrenal origin with a Weiss's score of 5. Six months later, the tumor relapsed, and he had a second surgery and was sent for hormone assessment. Clinical examination showed a skinny man with severe fatigue. He had no Cushingoid features. Gynecomastia and galactorrhea were absent. Penile length, testicular volume, and body hair growth were normal. Several cutaneous nodules were present. Biological assessment showed high morning plasma cortisol, which failed to be suppressed by treatment with 2 mg dexamethasone. Plasma estradiol and 17OH progesterone levels were high, but his testosterone levels were low. Radiological exploration showed numerous metastases: pleural, pulmonary, retroperitoneal, and abdominal. He was treated with classical chemotherapy, but he died four months after diagnosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5534236/

Record 1083

pmcid: PMC5367223

pmid: 28458479

doi: 10.4103/jmh.JMH_92_16

title: Gestational Gigantomastia: A Systematic Review of Case Reports.

authors

- Mangla M
- Singla D

journal: Journal of mid-life health

journalAbbr: J Midlife Health

year: 2017

pubdate: 2017 Jan-Mar

volume: 8

issue: 1

pages: 40-44

abstract: INTRODUCTION: Gestational gigantomastia (GG) is a rare disease characterized by diffuse, extreme, and incapacitating enlargement of one or both breasts during pregnancy. Although benign, it can lead to a great social, emotional, and physical disability. A good and complete knowledge regarding this rare but distressing clinical situation is a must among all practicing physicians especially obstetricians. MATERIALS AND METHODS: A systematic review of all the case reports and short case series, published in the English language in various databases in the last 40 years, i.e. 1976 to 2016 was carried out. The main aim was to provide a summary and critical analysis of all the data and evidence regarding GG published in recent years. RESULTS: After considering all inclusion and exclusion criteria, a total of fifty case reports were finally analyzed. The risk factors, geographical distribution, associated diseases, and the main treatment modalities used for GG are discussed in detail in this article. CONCLUSION: Multidisciplinary team effort in the form of obstetrician, plastic surgeon and anesthetist, and pediatrician is required for a successful fetomaternal outcome.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5367223/

Record 1084

pmcid: PMC5463295

pmid: 28008864

doi: 10.4274/jcrpe.3908

title: Hypogonadotropic Hypogonadism due to Novel FGFR1 Mutations.

authors

- Akkuş G
- Kotan LD
- Durmaz E
- Mengen E
- Turan İ
- Ulubay A
- Gürbüz F
- Yüksel B
- Tetiker T
- Topaloğlu AK

journal: Journal of clinical research in pediatric endocrinology

journalAbbr: J Clin Res Pediatr Endocrinol

year: 2017

pubdate: 2017 Jun 1

volume: 9

issue: 2

pages: 95-100

abstract: OBJECTIVE: The underlying genetic etiology of hypogonadotropic hypogonadism (HH) is heterogeneous. Fibroblast growth factor signaling is pivotal in the ontogeny of gonadotropin-releasing hormone neurons. Loss-of-function mutations in FGFR1 gene cause variable HH phenotypes encompassing pubertal delay to idiopathic HH (IHH) or Kallmann syndrome (KS). As FGFR1 mutations are common, recognizing mutations and associated phenotypes may enhance clinical management. METHODS: Using a candidate gene approach, we screened 52 IHH/KS patients. RESULTS: We identified three novel (IVS3-1G>C and p.W2X, p.R209C) FGFR1 gene mutations. Despite predictive null protein function, patients from the novel mutation families had normosmic IHH without non-reproductive phenotype. CONCLUSION: These findings further emphasize the great variability of FGFR1 mutation phenotypes in IHH/KS.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5463295/

Record 1085

pmcid: PMC5686882

pmid: 29164157

doi: 10.1177/2324709617740905

title: Inflammatory Breast Cancer and Warm Antibody Autoimmune Hemolytic Anemia: A Rare Paraneoplastic Syndrome.

authors

- Ugoeke N
- Onweni C
- Treece J
- Pai V
- Arikapudi S
- Kulbacki E
- Bajaj K

journal: Journal of investigative medicine high impact case reports

journalAbbr: J Investig Med High Impact Case Rep

year: 2017

pubdate: 2017 Oct-Dec

volume: 5

issue: 4

pages: 2324709617740905

abstract: Autoimmune hemolytic anemia (AIHA) is a disease process that involves the destruction of red blood cells mediated by the humoral immune system. It can be characterized as a cold agglutinin syndrome, paroxysmal cold hemoglobinuria, and warm, mixed type, and drug-induced AIHA. Although a well-established relationship exists between the presence of AIHA and lymphoproliferative malignancy, AIHA rarely presents in association with solid malignancies. An analysis of the limited number of published cases of AIHA in association

with solid malignancies performed showed that AIHA may present before the diagnosis of a solid malignancy, concurrently with the presence of a solid malignancy, or even on resolution of a solid malignancy. Few cases of solid cancers associated with AIHA have been reported. AIHA rarely presents as a paraneoplastic syndrome indicating existence of a solid cancer. We report a case of inflammatory breast cancer with AIHA.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5686882/

Record 1086

pmcid: PMC5398431

pmid: 28403136

doi: 10.12659/msm.900271

title: Influence of CYP2C9 and COX-2 Genetic Polymorphisms on Clinical Efficacy of Non-Steroidal Anti-Inflammatory Drugs in Treatment of Ankylosing Spondylitis.

authors

- Wang Y
- Yi XD
- Lu HL

journal: Medical science monitor : international medical journal of experimental and clinical research

journalAbbr: Med Sci Monit

year: 2017

pubdate: 2017 Apr 12

volume: 23

issue:

pages: 1775-1782

abstract: BACKGROUND The aim of this study was to evaluate the relationships of CYP2C9 and COX-2 genetic polymorphisms with therapeutic efficacy of non-steroidal anti-inflammatory drugs (NSAIDs) in treatment of ankylosing spondylitis (AS). MATERIAL AND METHODS We enrolled 130 AS inpatients and outpatients in the Arthritis and Rheumatism Department of Peking University First Hospital and 106 healthy people getting routine check-ups between September 2013 and July 2014. CYP2C9 and COX-2 genetic polymorphisms were detected by PCR-RFLP. All AS patients underwent medical treatment and 12-week follow-up treatment. Score differences of BASDAI, ASAS20, ASAS50, and ASAS70 for AS patients with different genotypes before and after treatment were compared. RESULTS In terms of COX-2-1290A/G and -1195G/A gene polymorphism genotype and allele frequency, the case group and control group were obviously different (all P<0.05), but CYP2C9*3 polymorphism genotype and allele frequency were not statistically different between the 2 groups (P>0.05). AS patients had improved BASDAI, ASAS20, ASAS50, and ASAS70 scores after they received NSAID treatment (all P<0.05). Furthermore, the efficacy of NSAID in treatment of AS and COX-2 gene -1290A/G and -1195G/A polymorphism were associated (all P<0.05), but it is not associated with CYP2C9 *3 polymorphism (all P>0.05). CONCLUSIONS COX-2-1290A/G and -1195G/A polymorphism may increase AS risk and they both can be considered as biological indicators for prediction of efficacy of NSAIDs in treatment of AS.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5398431/

Record 1087

pmcid: PMC5264352

pmid: 28232919

doi: 10.1159/000450617

title: Investigation of the Plausibility of 5-Alpha-Reductase Inhibitor Syndrome.

authors

- Fertig R
- Shapiro J
- Bergfeld W
- Tosti A

journal: Skin appendage disorders

journalAbbr: Skin Appendage Disord

year: 2017

pubdate: 2017 Jan

volume: 2

issue: 3-4

pages: 120-129

abstract: Postfinasteride syndrome (PFS) is a term recently coined to characterize a constellation of reported undesirable side effects described in postmarketing reports and small uncontrolled studies that developed during or after stopping finasteride treatment, and persisted after drug discontinuation. Symptoms included decreased libido, erectile dysfunction, sexual anhedonia, decreased sperm count, gynecomastia, skin changes, cognitive impairment, fatigue, anxiety, depression, and suicidal ideation. The aim of this study is to review the existing medical literature for evidence-based research of permanent sexual dysfunction and mood changes during treatment with 5-alpha-reductase inhibitors including finasteride and dutasteride.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5264352/

Record 1088

pmcid: PMC5306971

pmid: 28255494

doi: 10.1155/2017/3491012

title: Juvenile Breast Hypertrophy: A Successful Breast Reduction of 14.9% Body Weight without Recurrence in a 5-Year Follow-Up.

authors

- Hisham A
- Abd Latib M
- Basiron N

journal: Case reports in surgery

journalAbbr: Case Rep Surg

year: 2017

pubdate: 2017

volume: 2017

issue:

pages: 3491012

abstract: Juvenile hypertrophy of the breast (JHB) is a rare and relentless disease affecting women in the peripubertal period. We present a 13-year-old girl with massive bilateral JHB, successfully treated with a breast reduction and free nipple graft technique. A total of 7300 grams of breast tissue had been removed, accounting for 14.9% of the patient's total body weight. Prophylactic hormonal therapy was not commenced. During the 5-year follow-up period, there was no recurrence and the patient remains satisfied with the aesthetic outcome. A recent meta-analysis study indicates that subcutaneous mastectomy is associated with reduced risk of

recurrence, but it is more deforming and the aesthetic result is inferior to a reduction mammaplasty. In patients treated with the latter technique, some evidence exists suggesting that the use of a free nipple graft is associated with a less frequent risk of recurrence than a pedicle technique. This present case is unique as it demonstrates the clinical course of this patient at a considerably longer follow-up period than most reported studies. We adhered to the limited available evidence and highlight the long-term reliability of breast reduction with free nipple grafting as the first line surgical option in JHB, eliminating the need for repeated surgeries.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5306971/

Record 1089

pmcid: PMC5599879

pmid: 28911335

doi: 10.1186/s13256-017-1432-y

title: Mammary phyllodes tumor with six episodes of a relapse: a case report.

authors

- Iimori N
- Kashiwagi S
- Ishikawa T
- Kawajiri H
- Takashima T
- Ohsawa M
- Hirakawa K
- Ohira M

journal: Journal of medical case reports

journalAbbr: J Med Case Rep

year: 2017

pubdate: 2017 Sep 15

volume: 11

issue: 1

pages: 261

abstract: BACKGROUND: Phyllodes tumor is a rare breast mass. Most phyllodes tumors are benign, but occasionally some show malignancy. Even if the tumors are benign, they can easily show recurrence. CASE PRESENTATION: We report a case of a 48-year-old Asian woman, who had previously undergone a tumorectomy of her left breast 12 years before, with a pathological diagnosis of fibroadenoma. Five years after the initial tumorectomy, the patient presented with an abnormally enlarged left breast. A biopsy determined the growth to be a phyllodes tumor; subsequently, a partial mastectomy was conducted. However, the patient's left breast showed rapid enlargement in the next 5 months. The treating physicians suspected a relapse and subsequently consulted with our hospital. The breast mass was resected at our institution. After this surgery, the patient had repeated episodes of relapse and underwent four additional operations. Since then, the patient has not had any additional relapse so far. CONCLUSIONS: We present a case of a phyllodes tumor with multiple episodes of relapse. Although phyllodes tumors commonly show relapse, this case was unique because of the number of episodes of relapse. This case highlights the need to carry out tumorectomy with adequate margins with subsequent careful observation to check for relapse.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5599879/

Record 1090

pmcid: PMC6166145

pmid: 28845839

doi: 10.23750/abm.v88i2.6665

title: Management of Adolescent Gynecomastia: An Update.

authors

- Soliman AT
- De Sanctis V
- Yassin M

journal: Acta bio-medica : Atenei Parmensis

journalAbbr: Acta Biomed

year: 2017

pubdate: 2017 Aug 23

volume: 88

issue: 2

pages: 204-213

abstract: Gynecomastia refers to an enlargement of the male breast caused by benign proliferation of the glands ducts and stromal components including fat. It is the most common form of breast swelling seen in adolescent males. During pubertal development, gynecomastia can develop as a result of transient relative imbalances between androgens and estrogens. Pubertal gynecomastia is self-limited in 75 to 90% of adolescents and regresses over 1 to 3 years. However it may cause significant psychological stress and depression in adolescents. For boys with persistent gynecomastia that is causing substantial tenderness or embarrassment a tailored approach of close follow-up and use of anti-estrogen drugs may be recommended. These drugs block the effects of estrogens in the body and can reduce the size of the breasts somewhat. It appears that pharmacological therapy of persistent adolescent gynecomastia is reasonable effective if given early in the course of the disease and more successful in cases with small or moderate breast enlargement. However, neither of these drugs is universally approved for the treatment of gynecomastia because the risks and benefits have not been studied completely. Surgical approach may be needed under special conditions for cosmetic reasons. In this update, we review the different published trials for managing adolescent gynecomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6166145/

Record 1091

pmcid: PMC5506356

pmid: 28713712

doi:

title: Management of Gigantomastia: Outcomes of Superomedial Pedicle with Vertical Scar or Wise Pattern Skin Excision.

authors

- Sak MC
- Akın S
- Ersen B
- Tunalı O
- Ismail A

journal: World journal of plastic surgery

journalAbbr: World J Plast Surg

year: 2017

pubdate: 2017 May

volume: 6

issue: 2

pages: 206-211

abstract: BACKGROUND: Gigantomastia is a rare condition characterized by excessive breast growth and can be physically and psychosocially disabling for the patient. Regarding management of gigantomastia, this study evaluates the outcomes of superomedial pedicle with vertical scar or wise pattern skin excision. METHODS: A total of 425 patients who underwent reduction mammoplasty in our institution were reviewed. Forty eight reduction mammoplasty patients with resection weights greater than 1 kg per breast and treated with superomedial dermoglandular pedicle technique combined with vertical or wise-pattern skin excision were included. RESULTS: The patients were between 19 and 66 years old, with an average of 41 years. Total weight of resection was between 1000 and 2600 g, with an average of 1384 grams for right breast and between 1000 and 3000g, with an average of 1434 grams for left breast. The secondary revisions and wound healing complications were extremely high in vertical scar group compared to wise pattern group (87,5% and 12,5%, respectively). CONCLUSION: The authors concluded that superomedial dermoglandular pedicle in the addition of a wise pattern is an appropriate, safe and reliable method when dealing with significantly larger breasts (>1000g).

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5506356/

Record 1092

pmcid: PMC5406935

pmid: 28446213

doi: 10.1186/s12955-017-0655-3

title: Manifesto: towards a clinically-oriented psychometrics.

authors

- Vickers AJ
- Chen LY

journal: Health and quality of life outcomes

journalAbbr: Health Qual Life Outcomes

year: 2017

pubdate: 2017 Apr 26

volume: 15

issue: 1

pages: 83

abstract: BACKGROUND: New technologies to collect patient - reported outcomes have substantially solved the challenge of integrating a questionnaire in a busy clinical practice. At Memorial Sloan Kettering, we have been collecting patient reported outcomes electronically for many years. Our experience confirms the predicted benefits of obtaining patient reported outcomes but has also raised serious concerns about whether instruments developed for the research setting are appropriate for routine clinical use. DISCUSSION: We summarize four principles for a clinically - relevant psychometrics. First, minimize patient burden: the use of a large number of items for a single domain may be of interest for research but additional items have little clinical utility. Secondly, use simplified language: patients who do not have good language skills are typically excluded from research studies but will nonetheless present in clinical practice. Third, avoid dumb questions: many questionnaire items are inappropriate when applied to a more general population. Fourth, what works for the group may not work for the individual: group level statistics used to validate survey instruments can obscure problems when applied to a subgroup of patients. CONCLUSION: There is a need for a clinically-oriented psychometrics to help design, test, and evaluate questionnaires that would be used in routine practice. Developing statistical methods to optimize

questionnaires will be highly challenging but needed to bring the potential of patient reported outcomes into widespread clinical use.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5406935/

Record 1093

pmcid: PMC5331476

pmid: 28293638

doi: 10.1155/2017/7481597

title: Metabolic Disorders in HIV-Infected Adolescents Receiving Protease Inhibitors.

authors

- Santiprabhob J
- Tanchaweng S
- Maturapat S
- Maleesatharn A
- Lermankul W
- Sricharoenchai S
- Wittawatmongkol O
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- Phongsamart W
- Chokephaibulkit K

journal: BioMed research international

journalAbbr: Biomed Res Int

year: 2017

pubdate: 2017

volume: 2017

issue:

pages: 7481597

abstract: Protease inhibitor (PI) may cause abnormal glucose metabolism, abnormal lipid metabolism, and metabolic syndrome in HIV-infected adults but less well studied in Asian adolescents. This cross-sectional study evaluated anthropometric factors, oral glucose tolerance test, and lipid profiles of perinatally HIV-infected Thai adolescents who had received PI-based antiretroviral therapy for at least 6 months. Eighty adolescents were enrolled [median (IQR) age 16.7 (14.6-18.0) years, 42 males]. Metabolic syndrome, prediabetes, and type 2 diabetes mellitus (T2DM) were found in 8 (10%), 17 (22.1%), and 3 (3.8%) adolescents, respectively. Dyslipidemia was found in 56 (70%) adolescents, with hypertriglyceridemia being the most common type. In multivariate analysis, presence of lipohypertrophy (OR: 25.7, 95% CI: 3.2-202.8; p = 0.002) and longer duration of PI use (OR: 1.04, 95% CI: 1.00-1.08; p = 0.023) were associated with metabolic syndrome. Obesity (OR: 7.71, 95% CI: 1.36-43.7; p = 0.021), presence of lipohypertrophy (OR: 62.9, 95% CI: 4.97-795.6; p = 0.001), and exposure to stavudine for ≥6 months (OR: 8.18, 95% CI: 1.37-48.7; p = 0.021) were associated with prediabetes/T2DM, while exposure to tenofovir for ≥6 months reduced the risk (OR: 0.17, 95% CI: 0.04-0.78; p = 0.022). Metabolic disorders were commonly found in adolescents receiving PI. Careful monitoring and early intervention to modify cardiovascular risk should be systematically implemented in this population particularly those with exposure to stavudine.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5331476/

Record 1094

pmcid: PMC6062382

pmid: 30090542

doi: 10.1039/c7tx00184c

title: Modulation of estrogen synthesis and metabolism by phytoestrogens in vitro and the implications for women's health.

authors

- van Duursen MBM

journal: Toxicology research

journalAbbr: Toxicol Res (Camb)

year: 2017

pubdate: 2017 Nov 1

volume: 6

issue: 6

pages: 772-794

abstract: Phytoestrogens are increasingly used as dietary supplements due to their suggested health promoting properties, but also by women for breast enhancement and relief of menopausal symptoms. Generally, phytoestrogens are considered to exert estrogenic activity via estrogen receptors (ERs), but they may also affect estrogen synthesis and metabolism locally in breast, endometrial and ovarian tissues. Considering that accurate regulation of local hormone levels is crucial for normal physiology, it is not surprising that interference with hormonal synthesis and metabolism is associated with a wide variety of women's health problems, varying from altered menstrual cycle to hormone-dependent cancers. Yet, studies on phytoestrogens have mainly focused on ER-mediated effects of soy-derived phytoestrogens, with less attention paid to steroid synthesis and metabolism or other phytoestrogens. This review aims to evaluate the potential of phytoestrogens to modulate local estrogen levels and the implications for women's health. For that, an overview is provided of the effects of commonly used phytoestrogens, i.e. 8-prenylnaringenin, biochanin A, daidzein, genistein, naringenin, resveratrol and quercetin, on estrogen synthesizing and metabolizing enzymes in vitro . The potential implications for women's health are assessed by comparing the in vitro effect concentrations with blood concentrations that can be found after intake of these phytoestrogens. Based on this evaluation, it can be concluded that high-dose supplements with phytoestrogens might affect breast and endometrial health or fertility in women via the modulation of steroid hormone levels. However, more data regarding the tissue levels of phytoestrogens and effect data from dedicated, tissue-specific assays are needed for a better understanding of potential risks. At least until more certainty regarding the safety has been established, especially young women would better avoid using supplements containing high doses of phytoestrogens.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6062382/

Record 1095

pmcid: PMC5652866

pmid: 29025782

doi: 10.1136/bcr-2017-221899

title: Olfactory bulb agenesis with normal sexual hormones.

authors

- Tunsuriyawong P
- Pongpirul K
- Chaisam T
- Prajuabpansri P

journal: BMJ case reports

journalAbbr: BMJ Case Rep

year: 2017

pubdate: 2017 Oct 11

volume: 2017

issue:

pages:

abstract: An 18-year-old Caucasian man presented with a lack of sense of surrounding smell. The problem was first noticed when a family member discussed the smell of the food, which he had no idea what it was. The patient had normal development and sexual function, no history of trauma, surgery, chemical exposure or infection. Physical examination revealed no significant abnormalities. Smell threshold test using phenyl-ethyl-alcohol revealed bilateral anosmia. MRI showed bilateral aplastic olfactory bulbs and tracts associated with absent cortical growth of the olfactory sulci and asymmetrical gyrus rectus. Circulating hormones including cortisol, growth hormone, insulin-like growth factor 1, adrenocorticotropic hormone, thyroid hormones, follicle-stimulating hormone, luteinizing hormone, prolactin and testosterone were within normal ranges. Doppler ultrasound showed normal testis with bilateral supratesticular varicoceles. Given the loss of warning smell sensation, counselling for daily living precautions especially those related to gas, fire and rotten food was given.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5652866/

Record 1096

pmcid: PMC5360829

pmid: 28155090

doi: 10.1007/s40257-016-0245-x

title: Oral Spironolactone for Acne Vulgaris in Adult Females: A Hybrid Systematic Review.

authors

- Layton AM
- Eady EA
- Whitehouse H
- Del Rosso JQ
- Fedorowicz Z
- van Zuuren EJ

journal: American journal of clinical dermatology

journalAbbr: Am J Clin Dermatol

year: 2017

pubdate: 2017 Apr

volume: 18

issue: 2

pages: 169-191

abstract: BACKGROUND: The management of acne in adult females is problematic, with many having a history of treatment failure and some having a predisposition to androgen excess. Alternatives to oral antibiotics and combined oral contraceptives (COCs) are required. OBJECTIVE: Our aim was to conduct a hybrid systematic review of the evidence for benefits and potential harms of oral spironolactone in the management of acne in adult females. METHODS: The review was conducted according to a previously published protocol. Three reviewers independently selected relevant studies from the search results, extracted data, assessed the risk of bias, and rated the quality of the evidence using the Grading of Recommendations Assessment, Development and Evaluation (GRADE) approach. RESULTS: Ten randomized controlled trials (RCTs) and 21 case series

were retrieved. All trials were assessed as being at a 'high risk' of bias, and the quality of evidence was rated as low or very low for all outcomes. Apart from one crossover trial that demonstrated statistical superiority of a 200 mg daily dose versus inflamed lesions compared with placebo, data from the remaining trials were unhelpful in establishing the degree of efficacy of lower doses versus active comparators or placebo. Menstrual side effects were significantly more common with the 200 mg dose; frequency could be significantly reduced by concomitant use of a COC. Pooling of results for serum potassium supported the recent recommendation that routine monitoring is not required in this patient population. CONCLUSION: This systematic review of RCTs and case series identified evidence of limited quality to underpin the expert endorsement of spironolactone at the doses typically used (≤100 mg/day) in everyday clinical practice.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5360829/

Record 1097

pmcid: PMC5562972

pmid: 28821302

doi: 10.1186/s12902-017-0198-y

title: Participation of adults with disorders/differences of sex development (DSD) in the clinical study dsd-LIFE: design, methodology, recruitment, data quality and study population.

authors

- Röhle R
- Gehrmann K
- Szarras-Czapnik M
- Claahsen-van der Grinten H
- Pienkowski C
- Bouvattier C
- Cohen-Kettenis P
- Nordenström A
- Thyen U
- Köhler B

journal: BMC endocrine disorders

journalAbbr: BMC Endocr Disord

year: 2017

pubdate: 2017 Aug 18

volume: 17

issue: 1

pages: 52

abstract: BACKGROUND: dsd-LIFE is a comprehensive cross-sectional clinical outcome study of individuals with disorders/differences of sex development (DSD). This study focuses on various rare genetic conditions characterized by impaired gonadal or adrenal functionality. METHODS/DESIGN: The study aims to assess quality of life (QoL) as a measure of psychosocial adaptation, psychosexual and mental health aspects as major outcomes. Health status and functioning, medical and surgical therapies, participants' views on health care, psychological and social support, sociodemographic factors and their interrelations will be investigated as factors associated with the outcomes. In addition, ethical considerations in the field of DSD are addressed and previous experiences with health care were gathered. One thousand and forty participants with different DSD conditions were recruited by 14 study centres in 6 European countries (France, Germany, the Netherlands, Poland, Sweden and the United Kingdom) from February 2014 until September 2015. The conditions included were: Turner syndrome (n = 301); 45,X0/46,XY conditions (n = 45); Klinefelter syndrome (n = 218); 47,XYY (n =

1); 46,XY gonadal dysgenesis/ovotestes (n = 63); complete androgen insensitivity (CAIS) (n = 71); partial androgen insensitivity (PAIS) (n = 35) and androgen synthesis disorders (n = 20); severe hypospadias (n = 25); other or non-classified 46,XY DSD (n = 8); 46,XX congenital adrenal hyperplasia (CAH) (n = 226); 46,XX gonadal dysgenesis/ovotestis (n = 21); and 46,XX in males (n = 6). For an add-on study, 121 46,XY male-assigned individuals with CAH due to 21-hydroxylase deficiency were recruited. Mean age of participants' was 32.4 (+/- 13.6 years). DISCUSSION: Participation was high in conditions not commonly described as DSD, such as Turner and Klinefelter syndromes or CAH. Recruitment of individuals with XY DSD conditions proved to be more difficult. The data collection of PROs resulted in high data quality. Within medical and physical examination data, more missings and/or inaccurate data were found than expected. The European dsd-LIFE study recruited and evaluated the largest cross-sectional sample of individuals with different conditions classified under the term DSD. The data from this large sample will provide a sufficient basis for evidence-based recommendations for improvement of clinical care of individuals affected by a DSD condition. TRIAL REGISTRATION: German Clinical Trials Register DRKS00006072 .

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5562972/

Record 1098

pmcid: PMC5513689

pmid: 28988577

doi: 10.35946/arcr.v38.2.08

title: Pathophysiology of the Effects of Alcohol Abuse on the Endocrine System.

authors

- Rachdaoui N
- Sarkar DK

journal: Alcohol research : current reviews

journalAbbr: Alcohol Res

year: 2017

pubdate: 2017

volume: 38

issue: 2

pages: 255-276

abstract: Alcohol can permeate virtually every organ and tissue in the body, resulting in tissue injury and organ dysfunction. Considerable evidence indicates that alcohol abuse results in clinical abnormalities of one of the body's most important systems, the endocrine system. This system ensures proper communication between various organs, also interfacing with the immune and nervous systems, and is essential for maintaining a constant internal environment. The endocrine system includes the hypothalamic-pituitary-adrenal axis, the hypothalamic-pituitary-gonadal axis, the hypothalamic-pituitary-thyroid axis, the hypothalamic-pituitary-growth hormone/insulin-like growth factor-1 axis, and the hypothalamic-posterior pituitary axis, as well as other sources of hormones, such as the endocrine pancreas and endocrine adipose tissue. Alcohol abuse disrupts all of these systems and causes hormonal disturbances that may result in various disorders, such as stress intolerance, reproductive dysfunction, thyroid problems, immune abnormalities, and psychological and behavioral disorders. Studies in both humans and animal models have helped shed light on alcohol's effects on various components of the endocrine system and their consequences.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5513689/

Record 1099

pmcid: PMC5234247

pmid: 28086977

doi: 10.1186/s13104-017-2377-3

title: People living with HIV on ART have accurate perception of lipodystrophy signs: a cross-sectional study.

authors

- Alencastro PR
- Barcellos NT
- Wolff FH
- Ikeda ML
- Schuelter-Trevisol F
- Brandão AB
- Fuchs SC

journal: BMC research notes

journalAbbr: BMC Res Notes

year: 2017

pubdate: 2017 Jan 13

volume: 10

issue: 1

pages: 40

abstract: BACKGROUND: The prevalence of lipodystrophy ranges from 31 to 65%, depending on the criteria adopted for diagnosis. The usual methods applied in the diagnosis vary from self-perception, medical examination, skinfolds measurements, or even imaging assessment for confirmation of fat distribution changes. Although several methods have been developed, there is no gold standard for characterization of LA and LH, or mixed forms. This study aimed to compare self-reported signs of lipodystrophy with objective measures by skinfolds and circumferences, and to evaluate the prevalence of lipoatrophy (LA) and lipohypertrophy (LH) among subjects living with HIV/AIDS on ART. METHODS: A cross-sectional study enrolled participants living with HIV/AIDS receiving ART, aged 18 years or older from an outpatient health care center, in Southern Brazil. Self-reported body fat enlargement in the abdomen, chest or breasts, and dorsocervical fat pad were used to determine LH, while LA was identified by self-reported fat wasting of the face, neck, legs, arms or buttocks. Measurements were obtained with a scientific caliper for infraorbital, buccal, and submandibular skinfolds, and using an inelastic tape to measure circumferences of waist, hip, neck, and arm. LH and LA were established by the presence of at least one self-reported sign. RESULTS: Comparisons of self-reported signs with objective measurements for men and women were carried out in 815 participants on ART, out of 1240 participants with HIV infection. Self-report of decreased facial fat and sunken cheeks was associated with lower infraorbital, buccal, and submandibular skinfolds. Participants who reported buffalo hump had, on average, greater neck circumference, as well as those who have increased waist circumference also reported abdominal enlargement, but no buttock wasting. Men were most commonly affected by lipoatrophy (73 vs. 53%; $P < 0.001$), and women by lipohypertrophy (79 vs. 56%; $P < 0.001$). CONCLUSION: In conclusion, self-reported signs of lipodystrophy and lipoatrophy are prevalent, differ by gender, and are associated with objective measurements in people living with HIV/AIDS.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5234247/

Record 1100

pmcid: PMC5446543

pmid: 28560011

doi:

title: Peutz-Jeghers syndrome with early onset of pre-adolescent gynecomastia: a predigree case report and clinical and molecular genetic analysis.

authors

- Zhang LJ
- Su Z
- Liu X
- Wang L
- Zhang Q

journal: American journal of translational research

journalAbbr: Am J Transl Res

year: 2017

pubdate: 2017

volume: 9

issue: 5

pages: 2639-2644

abstract: This study reports a case of Peutz-Jeghers syndrome with early onset of gynecomastia, and discusses its clinical characteristics and genetic changes in a family. The clinical characteristics of a child diagnosed with Peutz-Jeghers syndrome in our hospital and his parents were summarized, and related genes were detected in the child and his parents. Furthermore, the therapeutic effect of letrozole was also observed. A five-year-and-three-month-old male patient visited a doctor due to "progressive painless enlargements at bilateral breast for more than two years". The mother of the patient had breast hyperplasia and ovarian cysts, had no hematemesis and hematochezia history, and had two 1-mm pigmented spots on the palm side of the left thumb. The father of the patient revealed no abnormalities. In the child, luteinizing hormone (LH) release peak induced by luteinizing hormone releasing hormone (LHRH) excitation testing was 0.29 U/l, and follicle stimulating hormone (FSH) peak was 0.41 U/l. Karyotype: 46, XY. Gene sequencing revealed a mutation c.658C>T in the serine threonine kinase 11 (STK11) gene in the child and this mother, while the child's father was normal. After one year of oral administration of letrozole, the boy's breasts reduced to stage B2, bone age was 10 years and eight months old, and ΔBA/ΔCA ratio was <1. The patient had early onset of pre-adolescent gynecomastia, had no obvious gastrointestinal symptoms, presented with a few pigmented spots in the skin mucosa, and was diagnosed with Peutz-Jeghers syndrome by genetic testing. Letrozole treatment can effectively control the development of breast and progression of bone age.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5446543/

Record 1101

pmcid: PMC5463111

pmid: 28630768

doi: 10.1155/2017/9458741

title: Plastic Surgery Inclusion in the Undergraduate Medical Curriculum: Perception, Challenges, and Career Choice-A Comparative Study.

authors

- Farid M
- Vaughan R
- Thomas S

journal: Plastic surgery international

journalAbbr: Plast Surg Int

year: 2017

pubdate: 2017

volume: 2017

issue:

pages: 9458741

abstract: OBJECTIVE: The undergraduate medical curriculum has been overcrowded with core learning outcomes with no formal exposure to plastic surgery. The aim of this study was to compare medical students from two educational settings for the basic understanding, preferred learning method, and factors influencing a career choice in plastic surgery. DESIGN AND SETTING: A prospective cohort study based on a web-based anonymous questionnaire sent to final year medical students at Birmingham University (United Kingdom), McGill University (Canada), and a control group (non-medical staff). The questions were about plastic surgery: (1) source of information and basic understanding; (2) undergraduate curriculum inclusion and preferred learning methods; (3) factors influencing a career choice. A similar questionnaire was sent to non-medical staff (control group). The data was analysed based on categorical outcomes (Chi-square χ 2) and level of significance $p \leq 0.05$. RESULTS: Questionnaire was analysed for 243 students (Birmingham, n = 171/332, 52%) (McGill n = 72/132, 54%). Birmingham students (14%) considered the word "plastic" synonymous with "cosmetic" more than McGill students (4%, $p < 0.025$). Teaching was the main source of knowledge for McGill students (39%, $p < 0.001$) while Birmingham students and control group chose the media (70%, $p < 0.001$). McGill students (67%) more than Birmingham (49%, $p < 0.010$) considered curriculum inclusion. The preferred learning method was lectures for McGill students (61%, $p < 0.01$) but an optional module for Birmingham (61%). A similar proportion (18%) from both student groups considered a career in plastic surgery. CONCLUSIONS: Medical students recognised the need for plastic surgery inclusion in the undergraduate curriculum. There was a difference for plastic surgery source of information, operations, and preferred method of learning for students. The study highlighted the urgent need to reform plastic surgery undergraduate teaching in collaboration with national educational bodies worldwide.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5463111/

Record 1102

pmcid: PMC5415150

pmid: 28490965

doi: 10.1016/j.sjbs.2016.10.021

title: Protective effects of fennel oil extract against sodium valproate-induced hepatorenal damage in albino rats.

authors

- Al-Amoudi WM

journal: Saudi journal of biological sciences

journalAbbr: Saudi J Biol Sci

year: 2017

pubdate: 2017 May

volume: 24

issue: 4

pages: 915-924

abstract: Foeniculum vulgare (Apiaceae) is commonly known as fennel. This herb is well-known worldwide and traditionally used as curative herbal therapy for the treatment of epileptic disease, seizurescarminative, digestive, lactogogue, diuretic, treating respiratory and gastrointestinal disorders. The aim of present study is to investigate the possible effect of fennel oil against the toxicity of Sodium-Valproic (SVP) in albino rats. In order to assess the protection of fennel oil on SVP induced hepato- and nephro-toxicity, male albino rats were treated with 1 ml/kg b.w fennel oil 3 days/week for 6 weeks. The biochemical analyses of hepatic enzymes were evaluated by estimating blood biomarkers of liver and renal damage along with histological examination. The results obtained from this work showed that treating animals with SVP lead to many histopathological alterations in the liver and kidney tissues. The effect appeared in the liver tissue include leukocyte infiltrations, cytoplasmic

vacuolization of the hepatocytes, fatty degeneration and congestion of blood vessels. This commonly used chemical (SVP) caused some unwanted effects on the kidney cortex which histologically observed as degeneration in renal tubules, atrophy of the glomeruli and edema. Biochemical results also revealed an abnormal increase in the enzyme level of AST, SAT, ALP, bilirubin, creatinine and urea-nitrogen, with a noticed decrease in total protein content. However, the results of treated rats with SVP plus fennel oil showed some positive histopathological changes in both the liver and kidney tissues. These results have confirmed that fennel oil has positive effects on the histological structure of the liver and kidney and the biochemical levels of AST, ALT, ALP, bilirubin, total proteins, creatinine and urea. It is concluded that fennel oil has various pharmacological properties including antioxidant, anti-cancer activity, anti-inflammatory. These valuable effects might be due to the presence of aromatic compounds trans -anethole. This useful properties of fennel plant could be due to its antioxidant activity that prevents the toxicity of SVP.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5415150/

Record 1103

pmcid: PMC5522831

pmid: 28900530

doi:

title: Pseudoangiomatous stromal hyperplasia presenting as accessory axillary breast tissue.

authors

- Johnson HM
- Reisler T

journal: Eplasty

journalAbbr: Eplasty

year: 2017

pubdate: 2017

volume: 17

issue:

pages: ic19

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5522831/

Record 1104

pmcid: PMC5648016

pmid: 28571891

doi: 10.1016/j.phrs.2017.05.020

title: Renin angiotensin aldosterone inhibition in the treatment of cardiovascular disease.

authors

- Ferrario CM
- Mullick AE

journal: Pharmacological research

journalAbbr: Pharmacol Res

year: 2017

pubdate: 2017 Nov

volume: 125

issue: Pt A

pages: 57-71

abstract: A collective century of discoveries establishes the importance of the renin angiotensin aldosterone system in maintaining blood pressure, fluid volume and electrolyte homeostasis via autocrine, paracrine and endocrine signaling. While research continues to yield new functions of angiotensin II and angiotensin-(1-7), the gap between basic research and clinical application of these new findings is widening. As data accumulates on the efficacy of angiotensin converting enzyme inhibitors and angiotensin II receptor blockers as drugs of fundamental importance in the treatment of cardiovascular and renal disorders, it is becoming apparent that the achieved clinical benefits is suboptimal and surprisingly no different than what can be achieved with other therapeutic interventions. We discuss this issue and summarize new pathways and mechanisms effecting the synthesis and actions of angiotensin II. The presence of renin-independent non-canonical pathways for angiotensin II production are largely unaffected by agents inhibiting renin angiotensin system activity. Hence, new efforts should be directed to develop drugs that can effectively block the synthesis and/or action of intracellular angiotensin II. Improved drug penetration into cardiac or renal sites of disease, inhibiting chymase the primary angiotensin II forming enzyme in the human heart, and/or inhibiting angiotensinogen synthesis would all be more effective strategies to inhibit the system. Additionally, given the role of angiotensin II in the maintenance of renal homeostatic mechanisms, any new inhibitor should possess greater selectivity of targeting pathogenic angiotensin II signaling processes and thereby limit inappropriate inhibition.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5648016/

Record 1105

pmcid: PMC5463291

pmid: 27873740

doi: 10.4274/jcrpe.3625

title: Response to Anastrozole Treatment in a Case with Peutz-Jeghers Syndrome and a Large Cell Calcifying Sertoli Cell Tumor.

authors

- Koç Yekedüz M
- Şıklar Z
- Burgu B
- Kuloğlu Z
- Kocaay P
- Çamtosun E
- İsakoca M
- Kansu A
- Soygür T
- Berberoğlu M

journal: Journal of clinical research in pediatric endocrinology

journalAbbr: J Clin Res Pediatr Endocrinol

year: 2017

pubdate: 2017 Jun 1

volume: 9

issue: 2

pages: 168-171

abstract: Peutz-Jeghers syndrome (PJS) is inherited as an autosomal dominant trait characterized by multiple gastrointestinal hamartomatous polyps, mucocutaneous pigmentation, and an increased risk of neoplasm. Large-cell calcifying Sertoli cell tumor (LCCSCT) is a kind of sex cord-stromal tumor which may co-exist with PJS and which is characterized radiologically by calcification foci within the testes. Surgical treatment options for this tumor range from testis-preserving surgery to radical orchiectomy. Not with standing this invasive approach, recently, there are some case reports demonstrating the efficacy of aromatase inhibitors in avoiding orchiectomy and its associated complications. In this paper, we have presented a LCCSCT case diagnosed in a boy with PJS and his response to anastrozole treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5463291/

Record 1106

pmcid: PMC5704513

pmid: 29204279

doi: 10.1530/EDM-17-0102

title: Sellar plasmacytoma presenting with symptoms of anterior pituitary dysfunction.

authors

- Ferreira AG
- Silva TN
- Luiz HV
- Campos FD
- Cordeiro MC
- Portugal JR

journal: Endocrinology, diabetes & metabolism case reports

journalAbbr: Endocrinol Diabetes Metab Case Rep

year: 2017

pubdate: 2017

volume: 2017

issue:

pages:

abstract: UNLABELLED: Sellar plasmacytomas are rare and the differential diagnosis with non-functioning pituitary adenomas might be difficult because of clinical and radiological resemblance. They usually present with neurological signs and intact anterior pituitary function. Some may already have or eventually progress to multiple myeloma. We describe a case associated with extensive anterior pituitary involvement, which is a rare form of presentation. A 68-year-old man was referred to our Endocrinology outpatient clinic due to gynecomastia, reduced libido and sexual impotence. Physical examination, breast ultrasound and mammography confirmed bilateral gynecomastia. Blood tests revealed slight hyperprolactinemia, low testosterone levels, low cortisol levels and central hypothyroidism. Sellar MRI showed a heterogeneous sellar mass (56 × 60 × 61 mm), initially suspected as an invasive macroadenoma. After correcting the pituitary deficits with hydrocortisone and levothyroxine, the patient underwent transsphenoidal surgery. Histological examination revealed a plasmacytoma and multiple myeloma was ruled out. The patient was unsuccessfully treated with radiation therapy (no tumor shrinkage). Myeloma ultimately developed, with several other similar lesions in different locations. The patient was started on chemotherapy, had a bone marrow transplant and is now stable (progression free) on lenalidomide and dexamethasone. The presenting symptoms and panhypopituitarism persisted, requiring chronic replacement treatment with levothyroxine, hydrocortisone and testosterone. LEARNING POINTS: Plasmacytomas, although rare, are a possible type of sellar masses, which have a completely different treatment approach, so it is important to make the correct diagnosis.Usually, they present with neurological signs and symptoms and a well-preserved pituitary function, but our case shows that anterior pituitary function can be severely compromised.Making a more extensive evaluation (clinical and biochemical)

might provide some clues to this diagnosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5704513/

Record 1107

pmcid: PMC5695607

pmid: 29149740

doi: 10.1016/j.ijscr.2017.11.009

title: Skin-reducing mastectomy and direct-to-implant reconstruction in giant phyllodes tumour of breast: case report.

authors

- Ciancio F
- Innocenti A
- Cagiano L
- Portincasa A
- Parisi D

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2017

pubdate: 2017

volume: 41

issue:

pages: 356-359

abstract: BACKGROUND: The phyllodes tumour of the breast comprises about 0.5% of all neoplasms of the breast. CASE REPORT: we present the case of a female patient (55 years old) suffering from giant phyllodes tumour (>10cm) of the left breast, who underwent Skin Reducing Mastectomy (SRM) and reconstruction with silicone breast implant (Bostwick Technique). DISCUSSION AND CONCLUSION: In selected patients with large sized breasts (C-D cup) and poor surface area of skin involved, a valid technique in the treatment of this pathology is the Skin-Reducing Mastectomy and immediate reconstruction with implants (DTI=direct to implant).

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5695607/

Record 1108

pmcid: PMC5440451

pmid: 28560306

doi: 10.1016/j.ijwd.2016.12.002

title: Spironolactone for the treatment of acne in women, a retrospective study of 110 patients.

authors

- Charny JW
- Choi JK
- James WD

journal: International journal of women's dermatology

journalAbbr: Int J Womens Dermatol

year: 2017

pubdate: 2017 Jun

volume: 3

issue: 2

pages: 111-115

abstract: BACKGROUND: There is limited evidence on the safety and efficacy of spironolactone in the treatment of women with acne. Thus, for many dermatologists spironolactone remains an alternative rather than a mainstay treatment for female patients with acne. METHODS: An electronic medical records search tool was used to select data from a group of women who received spironolactone to treat acne and were evaluated with the comprehensive acne severity scale (CASS) before treatment and at all follow-up visits. Data points were collected for CASS scores at each follow-up visit, concurrent and previous treatments, and side effects. These data points were used to draw conclusions about the safety and efficacy of spironolactone in this patient population. RESULTS: There were 110 patients that met all eligibility requirements. Of these, 94 patients saw an improvement in their CASS score and 61 patients completely cleared their score to 0. There were 16 patients who did not improve and six who relapsed after initial improvement. The women saw an average improvement in their acne by 73.1% for the face, 75.9% for the chest, and 77.6% for the back. Fifty-one women experienced side effects, but only six found them bothersome enough to stop taking spironolactone. CONCLUSION: A majority of women in this study saw a dramatic improvement in their acne while treated with spironolactone. There were low rates of relapse or discontinuation of the medication. To further promote the use of spironolactone as a first-line systemic treatment for women with acne, there must be more prospective controlled trials.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5440451/

Record 1109

pmcid: PMC5333348

pmid:

doi: 10.1002/jmrs.2_214

title: Sunday 26 March 2017.

authors

journal: Journal of Medical Radiation Sciences

journalAbbr: J Med Radiat Sci

year: 2017

pubdate: 2017 Mar

volume: 64

issue: Suppl 1

pages: 62-82

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5333348/

Record 1110

pmcid: PMC5695797

pmid: 29155891

doi: 10.1371/journal.pone.0188379

title: The burden of gynecomastia among men on antiretroviral therapy in Zomba, Malawi.

authors

- Singano V
- Amberbir A
- Garone D
- Kandionamaso C
- Msonko J
- van Lettow M
- Kalima K
- Mataka Y
- Kawalazira G
- Mateyu G
- Kwekwesa A
- Matengeni A
- van Oosterhout JJ

journal: PloS one

journalAbbr: PLoS One

year: 2017

pubdate: 2017

volume: 12

issue: 11

pages: e0188379

abstract: BACKGROUND: Many Africans who are on life-saving ART face challenges from a variety of toxicities. After the introduction of a standardized first-line efavirenz-containing ART regimen, reports of gynecomastia appeared in Malawian popular media, however data on the prevalence and risk factors of gynecomastia from Africa are lacking. METHODS: We conducted a cross-sectional study in males ≥18 years registered on ART at the HIV clinic in Zomba Central Hospital. Men who reported to have ever experienced breast or nipple enlargement received a standard questionnaire and underwent physical examination. Questions included perceptions and concerns about gynecomastia. Clinicians confirmed the presence and severity of gynecomastia. Routinely collected data on current and previous ART regimens, CD4 count, WHO clinical stage, anthropometric measurements and history of tuberculosis were extracted from the electronic database. RESULTS: We enrolled 1,027 men with median age 44 years (IQR: 38-52). The median ART duration was 57 months (IQR: 27-85); 46.7% were in WHO stage III/IV at ART initiation, 88.2% had exposure to efavirenz and 9% were overweight or obese. The prevalence of self-reported gynecomastia was 6.0% (62/1027) (95%-CI: 4.7-7.7%). Of men with gynecomastia 83.6% reported nipple enlargement and 98.4% enlarged breasts (85.5% bilateral). One-third said they had not reported gynecomastia to a health care worker. Over three-quarters mentioned that gynecomastia was an important or very important problem for them, while more than half were embarrassed by it. On examination gynecomastia was present in 90% (confirmed gynecomastia prevalence 5.5%; 95%-CI: 4.2-7.0%) and 51.8% had severity grade III or IV. History of tuberculosis treatment was independently associated with self-reported gynecomastia, adjusted OR 2.10 (95%-CI: 1.04-4.25). CONCLUSIONS: The burden of gynecomastia among men on ART in Malawi was higher than previously reported, and was associated with adverse psychological consequences, calling for increased awareness, a proactive diagnostic approach and diligent clinical management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5695797/

Record 1111

pmcid: PMC5422692

pmid: 28540239

doi: 10.21037/tau.2017.03.57

title: The impact and management of sexual dysfunction secondary to pharmacological therapy of benign prostatic hyperplasia.

authors

- Gandhi J
- Weissbart SJ
- Smith NL
- Kaplan SA
- Dagur G
- Zumbo A
- Joshi G
- Khan SA

journal: Translational andrology and urology

journalAbbr: Transl Androl Urol

year: 2017

pubdate: 2017 Apr

volume: 6

issue: 2

pages: 295-304

abstract: Benign prostatic hyperplasia (BPH) is one of the most common genitourinary complications in men over 50 years of age and typically presents with lower urinary tract symptoms (LUTS). Classes of medications include α1-adrenoceptor blockers, 5α-reductase inhibitors, and phosphodiesterase 5 inhibitors. Today, α1-adrenoceptor blockers and 5α-reductase inhibitors are often combined to give a synergistic effect. A review of the current literature identified several adverse sexual side effects, including erectile dysfunction (ED), decreased libido, orgasmic disorders, and ejaculatory disorders. We believe it is important to know the extent of these side effects, as the clinician and patient will need to decide the cost of improved voiding symptoms. The chief adverse effect is ejaculatory disorders, including the absence of ejaculation. Clinical consideration for BPH should include the elements of male sexual function, patients' age, and the characteristics and comprehensive effects of each group of drugs. Methodological bias in clinical studies, such as the subjective evaluation of the sexual side effect, makes it difficult to determine the ideal drug for treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5422692/

Record 1112

pmcid: PMC5648284

pmid: 29049351

doi: 10.1371/journal.pone.0186591

title: The impact of breast reduction surgery on breastfeeding: Systematic review of observational studies.

authors

- Kraut RY
- Brown E
- Korownyk C
- Katz LS
- Vandermeer B
- Babenko O
- Gross MS

- Campbell S
- Allan GM

journal: PloS one

journalAbbr: PLoS One

year: 2017

pubdate: 2017

volume: 12

issue: 10

pages: e0186591

abstract: BACKGROUND: Almost half a million breast reduction surgeries are performed internationally each year, yet it is unclear how this type of surgery impacts breastfeeding. This is particularly important given the benefits of breastfeeding. OBJECTIVES: To determine if breast reduction surgery impacts breastfeeding success and whether different surgical techniques differentially impact breast feeding success. METHODS: Databases were searched up to September 5, 2017. Studies were included if they reported the number of women successful at breastfeeding or lactation after breast reduction surgery, and if they reported either the total number of women who had children following breast reduction surgery, or the total number of women who attempted to breastfeed following surgery. RESULTS: Of 1,212 studies, 51 studies met the inclusion criteria; they were located worldwide and had 31 distinct breast reduction techniques. The percentage of breastfeeding success among studies was highly variable. However, when analyzed by the preservation of the column of parenchyma from the nipple areola complex to the chest wall (subareolar parenchyma), a clear pattern emerged. The median breastfeeding success was 4% (interquartile range (IQR) 0-38%) for techniques with no preservation, compared to 75% (IQR 37-100%) for techniques with partial preservation and 100% (IQR 75-100%) for techniques with full preservation. CONCLUSIONS: Techniques that preserve the column of subareolar parenchyma appear to have a greater likelihood of successful breastfeeding. The preservation of the column of subareolar parenchyma should be disclosed to women prior to surgery. Guidelines on the best breast reduction techniques to be used in women of child bearing years may be advantageous to ensure women have the greatest potential for successful breastfeeding after breast reduction surgery.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5648284/

Record 1113

pmcid: PMC5327034

pmid: 28212067

doi: 10.1089/cap.2016.0081

title: The Treatment of Severe Childhood Aggression Study: 12 Weeks of Extended, Blinded Treatment in Clinical Responders.

authors

- Findling RL
- Townsend L
- Brown NV
- Arnold LE
- Gadow KD
- Kolko DJ
- McNamara NK
- Gary DS
- Kaplin DB
- Farmer CA

- Kipp H
- Williams C
- Butter EM
- Bukstein OG
- Rice R Jr
- Buchan-Page K
- Molina BS
- Aman MG

journal: Journal of child and adolescent psychopharmacology

journalAbbr: J Child Adolesc Psychopharmacol

year: 2017

pubdate: 2017 Feb

volume: 27

issue: 1

pages: 52-65

abstract: OBJECTIVES: Previous "Treatment of Severe Childhood Aggression" (TOSCA) reports demonstrated that many children with severe physical aggression and attention-deficit/hyperactivity disorder (ADHD) responded well to two randomized treatments (parent training [PT]+stimulant+placebo = Basic vs. PT+stimulant+risperidone = Augmented) for 9 weeks. An important clinical question is whether these favorable outcomes are maintained over longer times. METHODS: Clinical responders to the 9-week trial (n = 103/168), defined as Clinical Global Impressions (CGI)-Improvement of much/very much improved plus substantial reduction in parent ratings of disruptiveness, were followed another 12 weeks (21 weeks total) while remaining on blinded treatment. Outcome measures included Clinical Global Impressions scale, Nisonger Child Behavior Rating Form (NCBRF), other parent/teacher-rated scales, laboratory tests, clinician ratings of abnormal movement, and other adverse events (AEs). RESULTS: Parent ratings of problem behavior showed minimal worsening of behavior from end of the 9-week acute trial (expected from regression to the mean after selecting best responders), but outcomes at Extension endpoint were meaningfully improved compared with acute study baseline. As expected, outcomes for Basic and Augmented treatment did not differ among these children selected for good clinical response. During Extension, more Augmented subjects had elevated prolactin; there were no clinically confirmed cases of tardive dyskinesia. Delayed sleep onset was the most frequent Basic AE. We also conducted a last-observation-carried-forward analysis, which included both nonresponders and responders. We found that, at the end of Extension, Augmented subjects had more improvement than Basic subjects on the NCBRF Positive Social subscale ($p = 0.005$; $d = 0.44$), the Antisocial Behavior Scale Reactive Aggression subscale ($p = 0.03$; $d = 0.36$), and marginally so on the Disruptive Behavior Total subscale ($p = 0.058$; $d = 0.29$, the primary outcome). CONCLUSIONS: The medium-term outcomes were good for the participants in both treatment groups, perhaps because they were selected for good response. When nonresponders were included in ITT analyses, there was some indication that Augmented surpassed Basic treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5327034/

Record 1114

pmcid: PMC5554565

pmid: 28827979

doi: 10.1155/2017/7438326

title: Trigeminal Neuralgia, Glossopharyngeal Neuralgia, and Myofascial Pain Dysfunction Syndrome: An Update.

authors

- Khan M
- Nishi SE
- Hassan SN
- Islam MA
- Gan SH

journal: Pain research & management

journalAbbr: Pain Res Manag

year: 2017

pubdate: 2017

volume: 2017

issue:

pages: 7438326

abstract: Neuropathic pain is a common phenomenon that affects millions of people worldwide. Maxillofacial structures consist of various tissues that receive frequent stimulation during food digestion. The unique functions (masticatory process and facial expression) of the maxillofacial structure require the exquisite organization of both the peripheral and central nervous systems. Neuralgia is painful paroxysmal disorder of the head-neck region characterized by some commonly shared features such as the unilateral pain, transience and recurrence of attacks, and superficial and shock-like pain at a trigger point. These types of pain can be experienced after nerve injury or as a part of diseases that affect peripheral and central nerve function, or they can be psychological. Since the trigeminal and glossopharyngeal nerves innervate the oral structure, trigeminal and glossopharyngeal neuralgia are the most common syndromes following myofascial pain dysfunction syndrome. Nevertheless, misdiagnoses are common. The aim of this review is to discuss the currently available diagnostic procedures and treatment options for trigeminal neuralgia, glossopharyngeal neuralgia, and myofascial pain dysfunction syndrome.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5554565/

Record 1115

pmcid: PMC5544650

pmid: 28824259

doi: 10.1007/s12288-016-0762-z

title: Unilateral Gynaecomastia in a Young Man with Chronic Myeloid Leukemia.

authors

- Jain A
- Varma S
- Garg R
- Malhotra P

journal: Indian journal of hematology & blood transfusion : an official journal of Indian Society of Hematology and Blood Transfusion

journalAbbr: Indian J Hematol Blood Transfus

year: 2017

pubdate: 2017 Sep

volume: 33

issue: 3

pages: 448-450

abstract: Male reproductive issues are frequently overlooked in patients of chronic myeloid leukemia (CML) on imatinib therapy. Current article describes a young man with CML on imatinib mesylate since 13 years who presented to us with painful left sided breast swelling. Mammography and fine needle aspiration cytology confirmed the diagnosis of gynaecomastia and hormone profile revealed low testosterone levels. Gynaecomastia was attributed to imatinib related hypogonadism. Gynaecomastia improved after hormone replacement therapy. Need for long term monitoring of reproductive hormones in patients of CML on imatinib therapy is emphasized in this report.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5544650/

Record 1116

pmcid: PMC6913570

pmid: 31851753

doi: 10.1016/j.isjp.2017.08.003

title: Use of autologous fat grafting in reconstruction following mastectomy and breast conserving surgery: An updated systematic review protocol.

authors

- Agha RA
- Borrelli MR
- Dowlut N
- Butt MF
- Freeman S
- Orkar TE
- Orgill DP

journal: International journal of surgery protocols

journalAbbr: Int J Surg Protoc

year: 2017

pubdate: 2017

volume: 5

issue:

pages: 22-26

abstract: INTRODUCTION: Use of autologous fat grafting (AFG) for breast reconstructive surgery is gaining acceptance, but concerns regarding its efficacy and safety remain. We present a protocol for a systematic review that aims to update the findings since our previous systematic review on a number of outcomes of AFG. METHODS: The systematic review has been registered a priori (UIN: reviewregistry308). All study designs, including randomised controlled trials, cohort studies, case-controlled studies and case reports/series, reporting original data, on women undergoing AFG for breast reconstruction following mastectomy or breast conserving surgery, will be included. Six categorical outcomes will be assessed: oncological; clinical; aesthetic/functional; patient-reported; process; and radiological.The search strategy will be devised to investigate 'fat grafting and breast reconstruction'. Electronic databases will be searched, 01 April 2014 to 21 August 2017: PubMed, MEDLINE®, EMBASE, SCOPUS, CINAHL, PsychINFO, SciELO, The Cochrane Library, including the Cochrane Central Register of Controlled Trials (CENTRAL), Database of Abstracts of Reviews of Effect (DARE), the Cochrane Methodology Register, Health Technology Assessment Database, the NHS Economic Evaluation Databases and Cochrane Groups, ClinicalTrials.gov, Current Controlled Trials Database, the World Health Organisation (WHO) International Clinical Trials Registry Platform, UpToDate.com, NHS Evidence and the York Centre for Reviews and Dissemination. Grey literature will be searched. Two trained, independent teams will screen all titles and abstracts, and relevant full texts, for eligibility. Data will be extracted under standardised extraction fields into a preformatted database. ETHICS AND DISSEMINATION: The systematic review will be

published in a peer-reviewed journal and presented at national and international meetings within fields of plastic, reconstructive and aesthetic surgery, and surgical oncology. The work will be disseminated electronically and in print. Brief reports of the review and findings will be disseminated to interested parties through email and direct communication. The review aims to guide healthcare practice and policy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6913570/

Record 1117

pmcid: PMC5403655

pmid: 28450956

doi: 10.3892/etm.2017.4086

title: VanWyk-Grumbach syndrome in a male pediatric patient: A rare case report and literature review.

authors

- Zhang S
- Yang J
- Zheng R
- Jiang L
- Wei Y
- Liu G

journal: Experimental and therapeutic medicine

journalAbbr: Exp Ther Med

year: 2017

pubdate: 2017 Mar

volume: 13

issue: 3

pages: 1151-1154

abstract: Primary hypothyroidism usually leads to retardation of linear growth and delay or even arrest of puberty in juvenile patients. In rare cases, pediatric patients with hypothyroidism may present with signs of VanWyk-Grumbach's syndrome (VWGS), which includes juvenile hypothyroidism, delayed bone age and pseudoprecocious puberty. The present study reported on a rare case of VWGS and other complications, including obesity, short stature, hepatomegaly and pituitary hyperplasia. In addition, a comprehensive literature review was performed to illustrate the treatment and outcome of VWGS in pediatric patients. The present study contributed to the current knowledge regarding the diagnosis and treatment of VWGS in pediatric patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5403655/

Record 1118

pmcid: PMC5929399

pmid: 27879556

doi: 10.1097/INF.0000000000001424

title: Who Gets Severe Gynecomastia Among HIV-infected Children in the United Kingdom and Ireland?

authors

- Kenny J
- Doerholt K
- Gibb DM

- Judd A

journal: The Pediatric infectious disease journal

journalAbbr: Pediatr Infect Dis J

year: 2017

pubdate: 2017 Mar

volume: 36

issue: 3

pages: 307-310

abstract: There are few data on gynecomastia in HIV-infected children. Within the UK/Ireland's national cohort, 56 of 1873 (3%) HIV-infected children had gynecomastia, of which 10 (0.5%) were severe. All 10 had received antiretroviral therapy for a median of 27.5 (21, 42) months; 4 of 10 had received efavirenz, 7 of 10 and 6 of 10 had received stavudine and/or didanosine respectively. Five were nonreversible, despite changing antiretroviral therapy, and required breast reduction surgery.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5929399/

Record 1119

pmcid: PMC12769453

pmid: 27470633

doi: 10.3760/cma.j.issn.1007-3418.2016.04.014

title: [Wilson's disease misdiagnosed as "nephritis" : a report of one case].

authors

- Liu Y
- Peng ZT
- Zhou B

journal: Zhonghua gan zang bing za zhi = Zhonghua ganzangbing zazhi = Chinese journal of hepatology

journalAbbr: Zhonghua Gan Zang Bing Za Zhi

year: 2016

pubdate: 2016 Apr

volume: 24

issue: 4

pages: 310-1

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC12769453/

Record 1120

pmcid: PMC5124344

pmid: 27921004

doi:

title: A Rare Case of Gestational Gigantomastia with Hypercalcemia: The Challenges of Management and Follow up.

authors

- Moazzami B

- Chaichian S
- Farahvash MR
- Taheri S
- Ahmadi SA
- Mokhtari M
- Sheibani K

journal: Journal of reproduction & infertility

journalAbbr: J Reprod Infertil

year: 2016

pubdate: 2016 Oct-Dec

volume: 17

issue: 4

pages: 243-246

abstract: BACKGROUND: Gigantomastia is a breast disorder marked by exaggerated rapid growth of the breasts, generally bilaterally. Since this disorder is very rare and has been reported only in sparse case reports its etiology has yet to be fully established. Treatment is aimed at improving the clinical and psychological symptoms and reducing the treatment side effects; however, the best therapeutic option varies from case to case. CASE PRESENTATION: The present report described a case of gestational gigantomastia in a 30-year-old woman, gravida 2, parity 1, 17 week pregnant admitted to Pars Hospital, Tehran, Iran, on May 2014. The patient was admitted to hospital at week 17 of pregnancy, although her breasts initially had begun to enlarge from the first trimester. The patient developed hypercalcemia in her 32nd week of pregnancy. The present report followed this patient from diagnosis until the completion of treatment. CONCLUSION: Although gestational gigantomastia is a rare condition, its timely prognosis and careful examination of some conditions like hyperprolactinemia and hypercalcemia is essential in successful management of this condition.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5124344/

Record 1121

pmcid: PMC4860650

pmid: 27070206

doi: 10.1089/bfm.2016.29008.pjb

title: ABM Clinical Protocol #20: Engorgement, Revised 2016.

authors

- Berens P
- Brodribb W

journal: Breastfeeding medicine : the official journal of the Academy of Breastfeeding Medicine

journalAbbr: Breastfeed Med

year: 2016

pubdate: 2016 May

volume: 11

issue: 4

pages: 159-63

abstract: A central goal of The Academy of Breastfeeding Medicine is the development of clinical protocols for managing common medical problems that may impact breastfeeding success. These protocols serve only as guidelines for the care of breastfeeding mothers and infants and do not delineate an exclusive course of

treatment or serve as standards of medical care. Variations in treatment may be appropriate according to the needs of an individual patient.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4860650/

Record 1122

pmcid: PMC5012631

pmid: 27511721

doi: 10.1200/JOP.2016.014241

title: Acute, Unilateral Breast Toxicity From Gemcitabine in the Setting of Thoracic Inlet Obstruction.

authors

- Weiskopf K
- Creighton D
- Lew T
- Caswell JL
- Ouyang D
- Shah AT
- Hofmann LV
- Neal JW
- Telli ML

journal: Journal of oncology practice

journalAbbr: J Oncol Pract

year: 2016

pubdate: 2016 Aug

volume: 12

issue: 8

pages: 763-4

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5012631/

Record 1123

pmcid: PMC5168892

pmid: 28003691

doi: 10.4103/0019-5049.195482

title: All India Difficult Airway Association 2016 guidelines for the management of unanticipated difficult tracheal intubation in obstetrics.

authors

- Ramkumar V
- Dinesh E
- Shetty SR
- Shah A
- Kundra P
- Das S

- Myatra SN
- Ahmed SM
- Divatia JV
- Patwa A
- Garg R
- Raveendra US
- Doctor JR
- Pawar DK
- Ramesh S

journal: Indian journal of anaesthesia

journalAbbr: Indian J Anaesth

year: 2016

pubdate: 2016 Dec

volume: 60

issue: 12

pages: 899-905

abstract: The various physiological changes in pregnancy make the parturient vulnerable for early and rapid desaturation. Severe hypoxaemia during intubation can potentially compromise two lives (mother and foetus). Thus tracheal intubation in the pregnant patient poses unique challenges, and necessitates meticulous planning, ready availability of equipment and expertise to ensure maternal and foetal safety. The All India Difficult Airway Association (AIDAA) proposes a stepwise plan for the safe management of the airway in obstetric patients. These guidelines have been developed based on available evidence; wherever robust evidence was lacking, recommendations were arrived at by consensus opinion of airway experts, incorporating the responses to a questionnaire sent to members of the AIDAA and the Indian Society of Anaesthesiologists (ISA). Modified rapid sequence induction using gentle intermittent positive pressure ventilation with pressure limited to ≤20 cm H 2 O is acceptable. Partial or complete release of cricoid pressure is recommended when face mask ventilation, placement of supraglottic airway device (SAD) or tracheal intubation prove difficult. One should call for early expert assistance. Maternal SpO 2 should be maintained ≥95%. Apnoeic oxygenation with nasal insufflation of 15 L/min oxygen during apnoea should be performed in all patients. If tracheal intubation fails, a second-generation SAD should be inserted. The decision to continue anaesthesia and surgery via the SAD, or perform fibreoptic-guided intubation via the SAD or wake up the patient depends on the urgency of surgery, foeto-maternal status and availability of resources and expertise. Emergency cricothyroidotomy must be performed if complete ventilation failure occurs.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5168892/

Record 1124

pmcid: PMC4925273

pmid: 27165168

doi: 10.1007/s10552-016-0758-x

title: Antidepressant use and circulating prolactin levels.

authors

- Reeves KW
- Okereke OI
- Qian J
- Tworoger SS

- Rice MS
- Hankinson SE

journal: Cancer causes & control : CCC

journalAbbr: Cancer Causes Control

year: 2016

pubdate: 2016 Jul

volume: 27

issue: 7

pages: 853-61

abstract: PURPOSE: To determine whether antidepressants (AD), specifically selective serotonin reuptake inhibitors (SSRIs), are linked to elevated prolactin levels among the general population. METHODS: Circulating prolactin levels were available for 4593 healthy participants in the Nurses' Health Study (NHS) and NHS2, including 267 AD users. We fit generalized linear models to calculate and compare adjusted mean prolactin levels between AD users and non-users and further among SSRI users. Multivariable logistic regression was used to estimate odds ratios (ORs) and 95 % confidence intervals (CIs) for "elevated" prolactin levels (>11 ng/mL) comparing AD users to non-users. We evaluated AD use and change in prolactin levels among 610 NHS participants with two measurements an average of 11 years apart. RESULTS: Adjusted geometric mean prolactin levels were similar among SSRI users (13.2 ng/mL, 95 % CI 12.2-14.4), users of other classes of ADs (12.7 ng/mL, 95 % CI 11.0-14.6), and non-users (13.1 ng/mL, 95 % CI 12.8-13.4). Neither AD use (OR 1.17, 95 % CI 0.89-1.53) nor SSRI use (OR 0.95, 95 % CI 0.66-1.38) was associated with elevated prolactin levels. Change in prolactin levels was similar across women who started, stopped, consistently used, or never used ADs. CONCLUSIONS: This study does not support the hypothesis that AD use would influence breast cancer risk via altered prolactin levels. These results provide some evidence that use of ADs to treat depression or other conditions may not substantially increase prolactin levels in the majority of women.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4925273/

Record 1125

pmcid: PMC11639617

pmid: 27502882

doi: 10.4103/0189-6725.187812

title: Association between plasma kisspeptin levels and adolescent gynecomastia.

authors

- Aluclu MA
- Sen S
- Cevik M

journal: African journal of paediatric surgery : AJPS

journalAbbr: Afr J Paediatr Surg

year: 2016

pubdate: 2016 Jul-Sep

volume: 13

issue: 3

pages: 136-9

abstract: BACKGROUND: Gynecomastia is defined as benign proliferation of male breast glandular tissue. To date, the pathophysiology of adolescent gynecomastia (AG) remains unclear. Kisspeptin is a polypeptide that plays an important role in the regulation of the hypothalamic-pituitary-gonadal hormonal axis. In this study, we

investigated whether there is a relationship between kisspeptin and AG. MATERIALS AND METHODS: This study included 40 males between 9 and 18 years of age diagnosed with gynecomastia. The control group consisted of 30 young healthy males in the same age range. The participants were evaluated with respect to anthropometric measurements (age, height, body weight, body mass index, breast and pubic stages and testicular volume). The levels of kisspeptin, follicle-stimulating hormone, luteinizing hormone, estradiol (E2), testosterone (T), and ratio of E2 to T were measured in both groups. RESULTS: The mean age was 13.8 years. There were no differences between the groups in terms of anthropometric parameters, plasma gonadotropin levels, estrogen levels, and E2/T ($P > 0.05$). Plasma kisspeptin (0.77 and 0.54 ng/mL, $P < 0.05$) and T (253.9 ng/dL and 117.9 ng/dL) levels were significantly higher in the AG group than in the control group ($P < 0.001$). CONCLUSION: Kisspeptin levels are an important factor in AG.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11639617/

Record 1126

pmcid: PMC4943238

pmid: 27032719

doi: 10.1161/JAHA.115.002493

title: Barriers to Adoption of Mineralocorticoid Receptor Antagonists in Patients With Heart Failure: A Mixed-Methods Study.

authors

- Dev S
- Hoffman TK
- Kavalieratos D
- Heidenreich P
- Wu WC
- Schwenke DC
- Tracy SJ

journal: Journal of the American Heart Association

journalAbbr: J Am Heart Assoc

year: 2016

pubdate: 2016 Mar 31

volume: 5

issue: 3

pages: e002493

abstract: BACKGROUND: Mineralocorticoid receptor antagonists (MRAs) are the most underutilized pharmacotherapy for heart failure. Minimal data are available on the barriers to MRA adoption from the perspective of prescribing clinicians. METHODS AND RESULTS: A mixed-methods study consisting of a survey (n=50), focus groups (n=39), interviews (n=6) with clinicians at a single US Department of Veterans Affairs medical center served to ascertain barriers to optimal use of MRAs. Participants were drawn from 6 groups: cardiology providers, cardiology fellows, hospitalists, clinical pharmacists, internal medicine residents, and primary care providers. Qualitative data were iteratively coded with qualitative data analysis software. The survey response rate was 17.3%. Overall, 51% of survey respondents were unfamiliar with eplerenone, and 6% were unfamiliar with spironolactone. In addition, 30% of respondents reported that they would order a laboratory test >2 weeks after a new MRA prescription, although that is beyond the guideline recommendation. Most providers correctly identified New York Heart Association class 3 and 4 patients as MRA eligible, but only 42% identified class 2 patients as MRA eligible. Through analysis of focus groups, we identified 8 barriers to MRA use in 3 categories: patient-based barriers (concerns about polypharmacy and comorbidities, adverse effects, perceived patient nonadherence), provider-based barriers (unclear roles and responsibilities, coordination and

transitions of care, lack of experience or familiarity with MRAs), and system-based barriers (system overload and provider time constraints, lack of systematic follow-up procedures). CONCLUSIONS: Eight primary barriers to MRA adoption at the provider, patient, and health system levels were identified from the prescriber perspective. These barriers can inform the creation of multilevel interventions that will be required to close the gap in MRA adoption.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4943238/

Record 1127

pmcid: PMC4876166

pmid: 27218035

doi: 10.5999/aps.2016.43.3.299

title: Bilateral Atypical Ductal Hyperplasia with Microcalcifications in a Patient with Gynecomastia.

authors

- Vidal P
- Berner JE
- Klenner A
- Tapia Ó

journal: Archives of plastic surgery

journalAbbr: Arch Plast Surg

year: 2016

pubdate: 2016 May

volume: 43

issue: 3

pages: 299-301

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4876166/

Record 1128

pmcid: PMC4885261

pmid: 27151057

doi: 10.1136/bcr-2016-215082

title: Bilateral breast enlargement in a chronic alcoholic: do not miss Madelung's disease.

authors

- Ray S
- Chakraborty PP
- Pramanik S
- Chowdhury S

journal: BMJ case reports

journalAbbr: BMJ Case Rep

year: 2016

pubdate: 2016 May 5

volume: 2016

issue:

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4885261/

Record 1129

pmcid: PMC5123024

pmid: 27920630

doi: 10.1159/000450867

title: Bilateral Diffuse Pseudoangiomatous Stromal Hyperplasia (PASH) Causing Gigantomastia in a 33-Year-Old Pregnant Woman: Case Report.

authors

- Krawczyk N
- Fehm T
- Ruckhäberle E
- Mohrmann S
- Riemer J
- Braunstein S
- Hoffmann J

journal: Breast care (Basel, Switzerland)

journalAbbr: Breast Care (Basel)

year: 2016

pubdate: 2016 Oct

volume: 11

issue: 5

pages: 356-358

abstract: Pseudoangiomatous stromal hyperplasia (PASH) is a benign proliferation of mammary stroma mostly described as an incidental microscopic finding. Clinically, it can manifest as a palpable, well-circumscribed breast mass or in rare cases as a diffuse bilateral process causing massive breast enlargement. The most postulated theory for aetiology of this condition is hormonal stimulation of mammary myofibroblasts, particularly by progesterone. A definite diagnosis of PASH is based on typical pathological findings like stromal hyperplasia and empty slit-like channels positive for myofibroblastic and negative for endothelial markers. The main clinical differential diagnosis is a fibroadedoma or phylloid tumour, and histologically a low-grade angiosarcoma. There are less than 200 cases of tumorous PASH and less than 20 of diffuse PASH reported so far. Here we present a case report of huge diffuse PASH, that is, to our knowledge, the first in a pregnant woman.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5123024/

Record 1130

pmcid: PMC4971350

pmid: 27563567

doi: 10.21037/gs.2016.03.05

title: Bilateral giant juvenile fibroadenomas of the breasts-a rare indication for bilateral skin reducing mastectomy.

authors

- Laitano FF
- Neto FL
- Zerwes FP

journal: Gland surgery

journalAbbr: Gland Surg

year: 2016

pubdate: 2016 Aug

volume: 5

issue: 4

pages: 435-8

abstract: Fibroadenoma is the most common benign tumor of the breast but giant juvenile fibroadenoma represent only 0.5% of all fibroadenomas and when bilateral are much more rare. We describe the case of a 25 years old girl that presented with bilateral giant juvenile fibroadenomas and was treated by bilateral skin reducing mastectomy using the inferior dermal flap, implant, and free nipple graft.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4971350/

Record 1131

pmcid: PMC5102722

pmid: 27867677

doi: 10.1155/2016/9084820

title: Bilateral Simultaneous Pseudoangiomatous Stromal Hyperplasia of the Breasts and Axillae: Imaging Findings with Pathological and Clinical Correlation.

authors

- Alikhassi A
- Ensani F
- Omranipour R
- Abdollahi A

journal: Case reports in radiology

journalAbbr: Case Rep Radiol

year: 2016

pubdate: 2016

volume: 2016

issue:

pages: 9084820

abstract: Pseudoangiomatous stromal hyperplasia (PASH) of the breast is a pathology that is usually diagnosed by accident during pathological examination of other breast lesions. PASH is an uncommon and benign tumoral lesion of the mammary stroma that can be pathologically mistaken for other tumours, such as phyllodes, fibroadenoma, and sometimes even angiosarcoma. We report the case of a 45-year-old woman with complaints of huge bilateral breast enlargement. This is a rare case of PASH presenting with gigantomastia and involving bilateral breasts and axillae simultaneously. Mammography, ultrasonography, and MRI features are illustrated with histopathological correlation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5102722/

Record 1132

pmcid: PMC4785695

pmid:

doi:

title: Book of Abstracts for the 11(th) Congress of the Association of Surgeons of Nigeria, Yenagoa, July 2015.

authors

journal: Nigerian Journal of Surgery : Official Publication of the Nigerian Surgical Research Society

journalAbbr: Niger J Surg

year: 2016

pubdate: 2016 Jan-Jun

volume: 22

issue: 1

pages: 50-2

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4785695/

Record 1133

pmcid: PMC5553464

pmid: 28824986

doi: 10.1055/s-0036-1584165

title: Breast Augmentation by Water-Jet Assisted Autologous Fat Grafting: A Report of 300 Operations.

authors

- Muench DP

journal: Surgery journal (New York, N.Y.)

journalAbbr: Surg J (N Y)

year: 2016

pubdate: 2016 Apr

volume: 2

issue: 2

pages: e19-e30

abstract: Background The BEAULI -method (Breast Augmentation by Lipotransfer) is available for extraction and processing of large transplantable fat quantities. The aim of this work is to describe the surgical technique precisely and reproducibly and to provide an overview of the autologous fat transfer based on surgical experience. Method The author performed 300 autologous fat transplantations on 254 women between September 3, 2010, and May 13, 2015. Patients desiring moderate volume increase, fuller and firmer breasts, as well as an optimization of the silhouette, ideally with the concurrent desire of the correction of unwanted fat deposits, were selected. The fat was extracted via water-jet assisted liposuction (Body-jet, Human Med AG, Schwerin, Germany), and the fat cells were subsequently separated with the Lipocollector ® (Human Med AG, Schwerin, Germany). Results The results were assessed with a control exam and photo comparison and were based on the responses on a questionnaire. Overall, 35.9% of the patients defined the result as very good, 38.6% as good, 22.4% as satisfactory, and 3.1% as poor. Conclusion This study shows that the autologous fat cell transplantation into the female breast via water-jet assisted liposuction achieves a moderate and harmoniously appearing breast volume enlargement as well as contour improvement. Further studies with more

cases and longer observation periods over several years could contribute to improving the method of the autologous fat transfer regarding the grow-in rate, efficiency, and safety.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5553464/

Record 1134

pmcid: PMC5119698

pmid: 27734795

doi: 10.11138/gchir/2016.37.3.118

title: Breast US as primary imaging modality for diagnosing gynecomastia.

authors

- Telegrafo M
- Introna T
- Coi L
- Cornacchia I
- Rella L
- Stabile Ianora AA
- Angelelli G
- Moschetta M

journal: Il Giornale di chirurgia

journalAbbr: G Chir

year: 2016

pubdate: 2016 May-Jun

volume: 37

issue: 3

pages: 118-122

abstract: AIM: To assess the role of breast US in diagnosing and classifying gynecomastia as the primary imaging modality and to compare US findings and classification system with the mammographic ones. PATIENTS AND METHODS: 48 patients suspected of having gynecomastia underwent mammography and US. Two radiologists in consensus retrospectively evaluated mammograms and sonograms. Both US and mammographic images were evaluated categorizing gynecomastia into non-mass, nodular and flame shaped patterns. The two category assignations were compared in order to find any difference. The reference standard for both the classification systems was represented by the cytological examination in 18 out of 44 cases (41%) and the six-month US follow-up in the remaining cases. RESULTS: The US examination revealed pseudo-gynecomastia in 4/48 (8%) and true gynecomastia in the remaining 44 (92%). Gynecomastia was bilateral in 25/44 cases (57%) and unilateral in the remaining 19 (43%). The cases of true gynecomastia included non mass shape in 26/44 cases (59%), nodular shape in 12 (27%) and flame shape in 6 (14%). The mammographic examination revealed the same results as compared with US findings. 18/44 (41%) patients affected by nodular or dendritic gynecomastia underwent cytological examination confirming the presence of glandular tissue and the benign nature of the clinical condition. CONCLUSIONS: US could be proposed as the primary imaging tool for diagnosing and classifying gynecomastia, avoiding unnecessary Xray examinations or invasive procedures in case of diffuse gynecomastia. In case of nodular or dendritic patterns, biopsy remains mandatory for a definitive diagnosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5119698/

Record 1135

pmcid: PMC4972970

pmid: 27488410

doi: 10.1186/s12885-016-2623-4

title: Case report: small cell transformation and metastasis to the breast in a patient with lung adenocarcinoma following maintenance treatment with epidermal growth factor receptor tyrosine kinase inhibitors.

authors

- Lin Q
- Cai GP
- Yang KY
- Yang L
- Chen CS
- Li YP

journal: BMC cancer

journalAbbr: BMC Cancer

year: 2016

pubdate: 2016 Aug 3

volume: 16

issue:

pages: 593

abstract: BACKGROUND: Breast metastasis from lung cancer has been reported, but not from SCLC that is transformed from lung adenocarcinoma during maintenance treatment with epidermal growth factor receptor tyrosine kinase inhibitor (EGFR-TKI). Transformation to small cell lung cancer(SCLC), although uncommonly seen, has been associated with resistance to EGFR-TKI therapy in lung adenocarcinomas. CASE PRESENTATION: We describe a case of a 49-year-old man with lung adenocarcinoma harboring L858R point mutation at the exon 21 of the epidermal growth factor receptor (EGFR). During the maintenance treatment with EGFR-TKI, the patient presented with a right breast mass, which was accompanied by elevated serum neuron specific enolase (NSE) level. The histological examination of biopsies from the breast mass and enlarging lung mass revealed SCLC that was less sensitive to standard SCLC treatment. The breast tumor was positive for thyroid transcription factor-1 (TTF-1), consistent with a lung primary cancer. CONCLUSION: This is the first case report of small cell transformation and metastatic to the breast in a patient with lung adenocarcinoma following EGFR-TKI treatment. Repeat biopsy is important for evaluation of evolving genetic and histologic changes and selection of appropriate treatment. and serum NSE measurement may be useful for detection of small cell transformation in cases with resistance to EGFR-TKI therapy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4972970/

Record 1136

pmcid: PMC5111210

pmid: 27891161

doi: 10.1186/s12981-016-0121-z

title: Cases of antiretroviral-associated gynaecomastia reported to the National HIV & Tuberculosis Health Care Worker Hotline in South Africa.

authors

- Njuguna C
- Swart A
- Blockman M
- Maartens G

- Chisholm B
- Stewart A
- Uys A
- Cohen K

journal: AIDS research and therapy

journalAbbr: AIDS Res Ther

year: 2016

pubdate: 2016

volume: 13

issue:

pages: 40

abstract: BACKGROUND: Gynaecomastia is associated with exposure to antiretroviral therapy (ART), in particular efavirenz. There is limited data on clinical characteristics of patients with ART-associated gynaecomastia in resource-limited settings and little guidance on the optimal management of this adverse drug reaction (ADR). We describe the clinical characteristics, management and outcomes of gynaecomastia cases reported to the National HIV & Tuberculosis Health Care Worker Hotline in South Africa. METHODS: We identified all gynaecomastia cases in adolescent boys and men on ART reported to the hotline between June 2013 and July 2014. We collected follow up data telephonically at monthly intervals to document clinical management and outcomes. RESULTS: We received 51 reports of gynaecomastia between June 2013 and July 2014; 11% of the 475 patient-specific ADR queries to the hotline. All patients were on efavirenz-based ART. Mean age was 34 years (standard deviation 12) and seven were adolescents. The median onset of gynaecomastia was 15 months after efavirenz initiation (interquartile range 6-42). Gynaecomastia was bilateral in 29 patients (57%) and unilateral in 16 (31%). Serum testosterone was quantified in 25 of 35 patients with follow up data, and was low in 2 (8%). Efavirenz was replaced with an alternative antiretroviral in 29/35 patients (83%) and gynaecomastia improved in 20/29 (69%). CONCLUSIONS: Gynaecomastia was a frequently reported ADR in our setting, occurring with prolonged efavirenz exposure. Testosterone was low in the minority of tested cases. Most clinicians elected to switch patients off efavirenz, and gynaecomastia improved in the majority.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5111210/

Record 1137

pmcid: PMC5044906

pmid: 27661036

doi: 10.1097/MD.0000000000004899

title: Chemotherapy against cancer during pregnancy: A systematic review on neonatal outcomes.

authors

- Esposito S
- Tenconi R
- Preti V
- Groppali E
- Principi N

journal: Medicine

journalAbbr: Medicine (Baltimore)

year: 2016

pubdate: 2016 Sep

volume: 95

issue: 38

pages: e4899

abstract: BACKGROUND: The concomitant incidence of cancer and pregnancy has increased in recent years because of the increase in maternal age at the time of the 1st pregnancy. The diagnosis of cancer in a pregnant woman causes ethical and therapeutic problems for both the patient and the physician. The main aim of this paper is to describe the available evidence concerning the short- and long-term neonatal impact of chemotherapy given to pregnant women. METHODS: The relevant publications in English were identified by a systematic review of MEDLINE and PubMed for the last 15 years. The search strategy included "cancer[Title/Abstract] OR tumor[Title/Abstract] AND pregnancy[Title/Abstract] OR pregnant[Title/Abstract] AND embryo[Title/Abstract] or fetus[Title/Abstract] or neonate[Title/Abstract] or newborn[Title/Abstract] or pediatric[Title/Abstract] or child[Title/Abstract] AND English[lang]." RESULTS: An analysis of the literature showed that only the administration of chemotherapy during the embryonic stage of conceptus is dangerous and can lead to the termination of the pregnancy. When the disease is diagnosed in the 2nd or 3rd trimester of gestation or when it is possible to delay the initiation of chemotherapy beyond the 14th week, the risk of severe problems for the fetus are low, and pregnancy termination is not required. CONCLUSION: Data regarding the final outcome of children who have received in utero chemotherapy seem reassuring. Only the administration in the embryonal stage of conceptus is dangerous and can lead to the termination of pregnancy. When the disease is diagnosed in the 2nd or 3rd trimester of gestation or when it is possible to delay the initiation of chemotherapy beyond the 14th week, the risk of severe problems for the fetus are low and pregnancy termination is not needed. Increased knowledge of how to minimize the risks of chemotherapy can reduce improper management including unnecessary termination of pregnancy, delayed maternal treatment, and iatrogenic preterm delivery.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5044906/

Record 1138

pmcid: PMC7672659

pmid: 26072926

doi: 10.1111/cge.12623

title: Congenital generalized lipodystrophy: identification of novel variants and expansion of clinical spectrum.

authors

- Haghighi A
- Kavehmanesh Z
- Haghighi A
- Salehzadeh F
- Santos-Simarro F
- Van Maldergem L
- Cimbalistiene L
- Collins F
- Chopra M
- Al-Sinani S
- Dastmalchian S
- de Silva DC
- Bakhti H
- Garg A
- Hilbert P

journal: Clinical genetics

journalAbbr: Clin Genet

year: 2016

pubdate: 2016 Apr

volume: 89

issue: 4

pages: 434-441

abstract: Congenital generalized lipodystrophy (CGL) is an autosomal recessive disorder with two major subtypes. Variants in AGPAT2 result in CGL type 1 with milder manifestations, whereas BSCL2 variants cause CGL type 2 with more severe features. Muscle hypertrophy caused by lack of adipose tissue is present early in life in CGL patients. Our aim was to investigate 10 CGL patients from 7 different countries and report genotype-phenotype relationships. Genetic analysis identified disease-causing variants in AGPAT2 (five patients) and in BSCL2 (five patients), including three novel variants; c.134C>A (p.Ser45*), c.216C>G (p.Tyr72*) in AGPAT2 and c.458C>A (p.Ser153*) in BSCL2. We also report possible novel clinical features such as anemia, breast enlargement, steatorrhea, intraventricular hemorrhage and nephrolithiasis in CGL patients. Generalized lipodystrophy and muscular hypertrophy were the only features in all of our patients. Hepatomegaly was the second common feature. Some manifestations were exclusively noticed in our CGL2 patients; hypertrichosis, high-pitched voice and umbilical hernia. Bone cysts and history of seizures were noticed only in CGL1 patients. The findings of this study expand our knowledge of genotype-phenotype correlations in CGL patients. These results have important clinical applications in diagnosis and management of the CGL patients as well as in genetic counseling in families at-risk.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7672659/

Record 1139

pmcid: PMC4966278

pmid: 26902950

doi: 10.1093/ijnp/pyw018

title: Efficacy and Safety of Paliperidone Palmitate 3-Month Formulation for Patients with Schizophrenia: A Randomized, Multicenter, Double-Blind, Noninferiority Study.

authors

- Savitz AJ
- Xu H
- Gopal S
- Nuamah I
- Ravenstijn P
- Janik A
- Schotte A
- Hough D
- Fleischhacker WW

journal: The international journal of neuropsychopharmacology

journalAbbr: Int J Neuropsychopharmacol

year: 2016

pubdate: 2016 Jul

volume: 19

issue: 7

pages:

abstract: BACKGROUND: This double-blind, parallel-group, multicenter, phase-3 study was designed to test the noninferiority of paliperidone palmitate 3-month formulation (PP3M) to the currently marketed 1-month formulation (PP1M) in patients (age 18-70 years) with schizophrenia, previously stabilized on PP1M. METHODS: After screening (≤3 weeks) and a 17-week, flexible-dosed, open-label phase (PP1M: day 1 [150mg eq. deltoid], day 8 [100mg eq. deltoid.], weeks 5, 9, and 13 [50, 75, 100, or 150mg eq., deltoid/gluteal]), clinically stable patients were randomized (1:1) to PP3M (fixed-dose, 175, 263, 350, or 525mg eq. deltoid/gluteal) or PP1M (fixed-dose, 50, 75, 100, or 150mg eq. deltoid/gluteal) for a 48-week double-blind phase. RESULTS: Overall, 1016/1429 open-label patients entered the double-blind phase (PP3M: n=504; PP1M: n=512) and 842 completed it (including patients with relapse). PP3M was noninferior to PP1M: relapse rates were similar in both groups (PP3M: n=37, 8%; PP1M: n=45, 9%; difference in relapse-free rate: 1.2% [95% CI:-2.7%; 5.1%]) based on Kaplan-Meier estimates (primary efficacy). Secondary endpoint results (changes from double-blind baseline in positive and negative symptom score total and subscale scores, Clinical Global Impression-Severity, and Personal and Social Performance scores) were consistent with primary endpoint results. No clinically relevant differences were observed in pharmacokinetic exposures between PP3M and PP1M. Both groups had similar tolerability profiles; increased weight was the most common treatment-emergent adverse event (double-blind phase; 21% each). No new safety signals were detected. CONCLUSION: Taken together, PP3M with its 3-month dosing interval is a unique option for relapse prevention in schizophrenia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4966278/

Record 1140

pmcid: PMC4835556

pmid: 27104174

doi: 10.6065/apem.2016.21.1.15

title: Endocrine dysfunctions in children with Williams-Beuren syndrome.

authors

- Kim YM
- Cho JH
- Kang E
- Kim GH
- Seo EJ
- Lee BH
- Choi JH
- Yoo HW

journal: Annals of pediatric endocrinology & metabolism

journalAbbr: Ann Pediatr Endocrinol Metab

year: 2016

pubdate: 2016 Mar

volume: 21

issue: 1

pages: 15-20

abstract: PURPOSE: Williams-Beuren syndrome (WBS) is caused by a hemizygous microdeletion of chromosome 7q11.23 and is characterized by global cognitive impairment, dysmorphic facial features, and supravalvular aortic stenosis. Endocrine dysfunctions have been reported in patients with WBS. This study was performed to investigate the frequency, clinical features, and outcomes of endocrine dysfunctions in children with WBS. METHODS: One hundred two patients were included. The diagnosis was confirmed by chromosome analysis and fluorescent in situ hybridization. Medical charts were reviewed retrospectively to analyze endocrine

dysfunctions such as short stature, precocious puberty, thyroid dysfunctions, and hypocalcemia. RESULTS: The age at diagnosis was 3.7±4.4 years (one month to 19 years). Height- and weight-standard deviation score (SDS) were -1.1±1.1 and -1.4±1.4 at presentation, respectively. Short stature was found in 26 patients (28.3%) among those older than 2 years. Body mass index-SDS increased as the patients grew older (P<0.001). Two males and one female (2.9%) were diagnosed with central precocious puberty. Nine patients (8.8%) were diagnosed with primary hypothyroidism at age 4.0±4.3 years (one month to 12.1 years); their serum thyroid stimulating hormone and free T4 levels were 15.2±5.4 μU/mL and 1.2±0.2 ng/dL, respectively. Hypercalcemia was observed in 12 out of 55 patients under age 3 (22%) at the age of 14.3±6.6 months (7 to 28 months) with a mean serum calcium level of 13.1±2.1 mg/dL. CONCLUSION: Endocrine dysfunctions are not uncommon causes of morbidity in patients with WBS. The severity and outcomes of their endocrine manifestations were heterogeneous. Long-term follow-up is needed to predict the prognosis of endocrine features.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4835556/

Record 1141

pmcid: PMC4865724

pmid: 27174509

doi: 10.1038/srep25865

title: External Volume Expansion Modulates Vascular Growth and Functional Maturation in a Swine Model.

authors

- Kao HK
- Hsu HH
- Chuang WY
- Chen SC
- Chen B
- Wu SC
- Guo L

journal: Scientific reports

journalAbbr: Sci Rep

year: 2016

pubdate: 2016 May 13

volume: 6

issue:

pages: 25865

abstract: Despite increasing application of the pre-grafting expansion during autologous fat transplantation in breast reconstruction, little is known about its mechanism of action. To address that, ventral skins of miniature pigs were treated over a 10-day or 21-day period, with continuous suction at -50 mm Hg via a 7-cm diameter rubber-lined suction-cup device. Soft tissue thickness increased immediately after this external volume expansion (EVE) treatment, such increase completely disappeared by the next day. In the dermis and subcutaneous fat, the EVE treated groups showed significant increases in blood vessel density evident by CD31 staining as well as in vascular networks layered with smooth muscle cells when compared with the control group. This finding was corroborated by the increased percentage of endothelial cells present in the treatment groups. There was no significant difference in the percentages of proliferating basal keratinocytes or adipocytes, nor in epidermal thickness. Moreover, the EVE had no effect on proliferation or differentiation potential of adipose stem cells. Taken together, the major effects of EVE appeared to be vascular remodeling and maturation of functional blood vessels. This understanding may help clinicians optimize the vascularity of the recipient bed to further improve fat graft survival.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4865724/

Record 1142

pmcid: PMC4928456

pmid: 27386051

doi:

title: Finasteride in Hidradenitis Suppurativa: A "Male" Therapy for a Predominantly "Female" Disease.

authors

- Khandalavala BN
- Do MV

journal: The Journal of clinical and aesthetic dermatology

journalAbbr: J Clin Aesthet Dermatol

year: 2016

pubdate: 2016 Jun

volume: 9

issue: 6

pages: 44-50

abstract: OBJECTIVE: Hidradenitis suppurativa is associated with obesity and metabolic syndrome, and a hormonal component has been implicated. Finasteride is an anti-androgenic agent used for benign prostatic hypertrophy, androgenic alopecia, and, in females, hirsutism. Finasteride is an inhibitor of type II5 alpha-reductase that reduces dihydrotestosterone levels and appears to alter end-organ sensitivity of the folliculopilosebaceous unit. The objective is to review the use of finasteride for hidradenitis suppurativa. DESIGN: Review of the literature. SETTING: Clinical treatment of patients with hidradenitis suppurativa. Measurement/participants: Five publications described the use for hidradenitis suppurativa. Four global case reports cited 13 individual patients, four male and nine female. Females included three adolescent patients and a child aged seven with precocious puberty. In the United States, finasteride in obese male adults was mentioned to be helpful. RESULTS: Oral finasteride, as monotherapy or additional therapy was utilized for advanced hidradenitis suppurativa. The outcomes were largely favorable, with complete resolution in three patients. A latency period was evident in a majority. Limited, or continuous use for up to six years, was detailed. Response to reintroduction was successful. A benign safety profile with excellent tolerability was described. Teratogenicity of finasteride was addressed and contraception advocated in female patients. Sexual adverse effects were not ascertained. CONCLUSION: In hidradenitis suppurativa, finasteride could be considered in adults of both sexes as well as in select female children and adolescents, particularly those with concurrent metabolic and hormonal alterations present. Finasteride provides another highly effective, durable, relatively safe, and inexpensive option in the treatment of hidradenitis suppurativa.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4928456/

Record 1143

pmcid: PMC5056272

pmid: 27752383

doi: 10.1155/2016/9487616

title: Galactocele in a Male Infant with Transient Hyperprolactinaemia: An Extremely Rare Cause of Breast Enlargement in Children.

authors

- Lau CT
- Wong KK
- Tam P

journal: Case reports in pediatrics

journalAbbr: Case Rep Pediatr

year: 2016

pubdate: 2016

volume: 2016

issue:

pages: 9487616

abstract: Galactocele is a rare breast condition in infants. Here, we report a 16-month-old boy who developed progressive left breast enlargement. Ultrasonography and magnetic resonance imaging revealed a 4 cm cystic lesion at left breast. Hormonal assay showed transient hyperprolactinaemia with no known cause identified. Subsequently, galactocele was confirmed on histopathological examination after complete surgical excision. No recurrence was observed on regular follow-up.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5056272/

Record 1144

pmcid: PMC4852364

pmid: 27195157

doi: 10.1155/2016/3576024

title: Gigantomastia and Macroprolactinemia Responding to Cabergoline Treatment: A Case Report and Minireview of the Literature.

authors

- Dellal FD
- Ozdemir D
- Aydin C
- Kaya G
- Ersoy R
- Cakir B

journal: Case reports in endocrinology

journalAbbr: Case Rep Endocrinol

year: 2016

pubdate: 2016

volume: 2016

issue:

pages: 3576024

abstract: Background. Macroprolactinemia is defined as predominance of high molecular weight prolactin forms in the circulation. Although macroprolactin is considered as a biologically inactive molecule, some authorities suggest treatment in symptomatic cases. Gigantomastia is defined as excess breast tissue and most cases in the literature were treated by surgical intervention. Case. A 44-year-old woman was admitted to our clinic with gigantomastia and galactorrhea. The patient had a demand for surgical therapy. In laboratory examination, she had hyperprolactinemia and macroprolactinemia. Pituitary imaging revealed 6 mm microadenoma in right side of the hypophysis. Since she was symptomatic, cabergolin treatment was started. Macroprolactin became negative, breast circumference decreased significantly, and galactorrhea resolved after treatment. Conclusion. Gigantomastia might be the presenting symptom in patients with macroprolactinemia. In these patients medical treatment with cabergoline may be used initially as an alternative to surgical approach.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4852364/

Record 1145

pmcid: PMC5348613

pmid: 28321284

doi: 10.4314/mmj.v28i4.8

title: Gynaecomastia in two men on stable antiretroviral therapy who commenced treatment for tuberculosis.

authors

- Kratz JD
- El-Shazly AY
- Mambuque SG
- Demetria E
- Veldkamp P
- Anderson TS

journal: Malawi medical journal : the journal of Medical Association of Malawi

journalAbbr: Malawi Med J

year: 2016

pubdate: 2016 Dec

volume: 28

issue: 4

pages: 185-187

abstract: Gynaecomastia is a common clinical presentation that varies from benign presentations in stages of human development to hormonal pathology, mainly due to hepatic dysfunction, malignancy, and adverse pharmacologic effects. We describe the development of significant bilateral gynaecomastia after starting treatment for pulmonary tuberculosis (TB) in two males with WHO stage III Human Immunodeficiency Virus (HIV) infection on stable antiretroviral regimens. Emerging reports suggest that distinct hepatic impairment in efavirenz metabolism modulates oestrogenic activity, which may be potentiated by anti-tuberculosis therapy. Clinical application includes early recognition of efavirenz-induced gynaecomastia, especially after commencing tuberculosis treatment. To avoid decreased adherence resulting from the distressing side effect of gynecomastia, transition to an alternative ART regimen over the course of tuberculosis treatment should be considered.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5348613/

Record 1146

pmcid: PMC5137796

pmid: 27912889

doi: 10.1016/j.fertnstert.2016.09.046

title: Hormone replacement therapy in young women with primary ovarian insufficiency and early menopause.

authors

- Sullivan SD
- Sarrel PM
- Nelson LM

journal: Fertility and sterility

journalAbbr: Fertil Steril

year: 2016

pubdate: 2016 Dec

volume: 106

issue: 7

pages: 1588-1599

abstract: Primary ovarian insufficiency (POI) is a rare but important cause of ovarian hormone deficiency and infertility in women. In addition to causing infertility, POI is associated with multiple health risks, including bothersome menopausal symptoms, decreased bone density and increased risk of fractures, early progression of cardiovascular disease, psychologic impact that may include depression, anxiety, and decreased perceived psychosocial support, potential early decline in cognition, and dry eye syndrome. Appropriate hormone replacement therapy (HRT) to replace premenopausal levels of ovarian sex steroids is paramount to increasing quality of life for women with POI and ameliorating associated health risks. In this review, we discuss POI and complications associated with this disorder, as well as safe and effective HRT options. To decrease morbidity associated with POI, we recommend using HRT formulations that most closely mimic normal ovarian hormone production and continuing HRT until the normal age of natural menopause, ~50 years. We address special populations of women with POI, including women with Turner syndrome, women with increased risk of breast or ovarian cancer, women approaching the age of natural menopause, and breastfeeding women.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5137796/

Record 1147

pmcid: PMC6463977

pmid: 27755646

doi: 10.1002/14651858.CD011579.pub2

title: Hydroxyurea for reducing blood transfusion in non-transfusion dependent beta thalassaemias.

authors

- Foong WC
- Ho JJ
- Loh CK
- Viprakasit V

journal: The Cochrane database of systematic reviews

journalAbbr: Cochrane Database Syst Rev

year: 2016

pubdate: 2016 Oct 18

volume: 10

issue: 10

pages: CD011579

abstract: BACKGROUND: Non-transfusion dependent beta thalassaemia is a subset of inherited haemoglobin disorders characterised by reduced production of the beta globin chain of the haemoglobin molecule leading to anaemia of varying severity. Although blood transfusion is not a necessity for survival, it is required when episodes of chronic anaemia occur. This chronic anaemia can impair growth and affect quality of life. People with non-transfusion dependent beta thalassaemia suffer from iron overload due to their body's increased capability of absorbing iron from food sources. Iron overload becomes more pronounced in those requiring blood transfusion. People with a higher foetal haemoglobin level have been found to require fewer blood transfusions. Hydroxyurea has been used to increase foetal haemoglobin level; however, its efficacy in reducing

transfusion, chronic anaemia complications and its safety need to be established. OBJECTIVES: To assess the effectiveness, safety and appropriate dose regimen of hydroxyurea in people with non-transfusion dependent beta thalassaemia (haemoglobin E combined with beta thalassaemia and beta thalassaemia intermedia). SEARCH METHODS: We searched the Cochrane Cystic Fibrosis and Genetic Disorders Group's Haemoglobinopathies Trials Register, compiled from electronic database searches and handsearching of relevant journals. We also searched ongoing trials registries and the reference lists of relevant articles and reviews.Date of last search: 30 April 2016. SELECTION CRITERIA: Randomised or quasi-randomised controlled trials of hydroxyurea in people with non-transfusion dependent beta thalassaemia comparing hydroxyurea with placebo or standard treatment or comparing different doses of hydroxyurea. DATA COLLECTION AND ANALYSIS: Two authors independently applied the inclusion criteria in order to select trials for inclusion. Both authors assessed the risk of bias of trials and extracted the data. A third author verified these assessments. MAIN RESULTS: No trials comparing hydroxyurea with placebo or standard care were found. However, we included one randomised controlled trial (n = 61) comparing 20 mg/kg/day with 10 mg/kg/day of hydroxyurea for 24 weeks.Both haemoglobin and foetal haemoglobin levels were lower at 24 weeks in the 20 mg group compared with the 10 mg group, mean difference -2.39 (95% confidence interval - 2.8 to -1.98) and mean difference -1.5 (95% confidence interval -1.83 to -1.17), respectively. Major adverse effects were significantly more common in the 20 mg group, for neutropenia risk ratio 9.93 (95% confidence interval 1.34 to 73.97) and for thrombocytopenia risk ratio 3.68 (95% confidence interval 1.13 to 12.07). No difference was reported for minor adverse effects (gastrointestinal disturbances and raised liver enzymes). The effect of hydroxyurea on transfusion frequency was not reported.The overall quality for the outcomes reported was graded as very low mainly because the outcomes were derived from only one small study with an unclear method of allocation concealment. AUTHORS' CONCLUSIONS: There is no evidence from randomised controlled trials to show whether hydroxyurea has any effect compared with controls on the need for blood transfusion. Administration of 10 mg/kg/day compared to 20 mg/kg/day of hydroxyurea resulted in higher haemoglobin levels and seems safer with fewer adverse effects. It has not been reported whether hydroxyurea is capable of reducing the need for blood transfusion. Large well-designed randomised controlled trials with sufficient duration of follow up are recommended.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6463977/

Record 1148

pmcid: PMC4860874

pmid: 27151467

doi: 10.3238/arztebl.2016.0270

title: In Reply.

authors

- Gerber B
- Marx M
- Untch M
- Faridi A

journal: Deutsches Arzteblatt international

journalAbbr: Dtsch Arztebl Int

year: 2016

pubdate: 2016 Apr 15

volume: 113

issue: 15

pages: 270

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4860874/

Record 1149

pmcid: PMC5611694

pmid: 28959793

doi: 10.18869/acadpub.cmm.2.4.30

title: In vitro activity of kombucha tea ethyl acetate fraction against Malassezia species isolated from seborrhoeic dermatitis.

authors

- Mahmoudi E
- Saeidi M
- Marashi MA
- Moafi A
- Mahmoodi V
- Zeinolabedini Zamani M

journal: Current medical mycology

journalAbbr: Curr Med Mycol

year: 2016

pubdate: 2016 Dec

volume: 2

issue: 4

pages: 30-36

abstract: BACKGROUND AND PURPOSE: Seborrheic dermatitis is a chronic and recurrent superficial dermatitis in which Malassezia species play an important role. There are different Malassezia species, which have been recently reported to be resistant to common antifungals. Natural sources can be useful alternatives to reduce the emergence of this resistance. Kombucha tea is believed to have potential antimicrobial properties. Regarding this, the present study aimed to investigate the antifungal activity of Kombucha tea ethyl acetate fraction (KEAF) against Malassezia species obtained from the patients with seborrheic dermatitis. MATERIALS AND METHODS: A total of 23 clinical isolates were identified by direct microscopic examination and Tween assimilation, and then confirmed by DNA sequencing of ITS regions for Malassezia species. Kombucha tea was fractionated using ethyl acetate (1:2 v/v). The minimum inhibitory concentration (MIC) microdilution assay was used to evaluate the anti- Malssezia activity of KEAF at three concentrations of 10, 40, and 80 mg/mL. RESULTS: The results of the DNA sequence analysis indicated that M. furfur (39.13%) was the predominant species, followed by M. globosa (30.43%), M. sloofie (13.04%), M. sympodialis (13.04%), and M. restricta (4.34%), respectively. Furthermore, KEAF showed inhibitory activity against Malassezia species. Accordingly, KEAF had the lowest and highest MIC value against M. sloofie and M. restricta , respectively. Moreover, the inhibitory effect of the extract was equivalent to that of ketoconazole at 4.8 µg/mL. CONCLUSION: The findings of the current study highlighted the antifungal properties of KEAF. Therefore, this extract can be promoted as complementary medicine for the treatment of the infections caused by Malassezia .

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5611694/

Record 1150

pmcid: PMC5098945

pmid: 27707680

doi: 10.5152/dir.2016.15539

title: Incidentally detected breast lesions on chest CT with US correlation: a pictorial essay.

authors

- Son JH
- Jung HK
- Song JW
- Baek HJ
- Doo KW
- Kim W
- Kim YM
- Kim WW
- Lee JS
- Cho EY

journal: Diagnostic and interventional radiology (Ankara, Turkey)

journalAbbr: Diagn Interv Radiol

year: 2016

pubdate: 2016 Nov-Dec

volume: 22

issue: 6

pages: 514-518

abstract: With the increasing use of computed tomography (CT), incidental breast lesions are detected more frequently. When interpreting chest CT findings, it is important for radiologists to carefully review the breast to recognize any abnormal findings that could affect patient management. The purpose of this study is to discuss incidental breast lesions on chest CT with ultrasonography correlation that may be encountered in routine clinical practice.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5098945/

Record 1151

pmcid: PMC4855789

pmid: 27132043

doi: 10.1016/j.ijscr.2016.04.031

title: Innovative combination of therapeutic mammoplasty and expandable-implant breast augmentation for immediate partial breast reconstruction.

authors

- Choo AM
- Forouhi P
- Malata CM

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2016

pubdate: 2016

volume: 23

issue:

pages: 146-50

abstract: INTRODUCTION: Therapeutic mammoplasty is used in the treatment of suitably-sized and

appropriately-located breast cancers to achieve adequate cancer excision, resulting in well-shaped but smaller breasts. In patients wishing to maintain or increase their breast size, simultaneous augmentation will be required. PRESENTATION OF CASE: A 48-year-old female underwent an "augmentation-therapeutic mastopexy". She required mastectomy for a multifocal cancer of the right breast and breast conservation for a unifocal localised cancer in the upper part of the left breast. She requested right immediate breast reconstruction and hoped for larger breasts than she had. Due to complications during neoadjuvant chemotherapy, the right reconstruction plan was changed from a deep inferior epigastric perforator (DIEP) flap to an implant-based technique. On the left, an extended superomedial pedicle therapeutic mammoplasty was combined with a subpectoral augmentation using an expandable-implant. DISCUSSION: The use of expandable-implants for reconstruction of partial mastectomy defects in combination with therapeutic mammoplasty has not been reported. This case report shows that such "augmentation-therapeutic mastopexy" is feasible. CONCLUSION: A "novel" oncoplastic technique herein termed "augmentation-therapeutic mastopexy" is described for partial breast reconstruction during the treatment of a patient with bilateral breast cancer. It enabled adequate treatment of her cancer while reshaping the breast and achieving the desired larger breast size. It should be considered in selected breast-conservation patients who wish to maintain or increase their breast size.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4855789/

Record 1152

pmcid: PMC4815277

pmid: 27047664

doi: 10.1530/EDM-16-0006

title: Juvenile granulosa cell tumour of the ovary presenting with hyperprolactinaemic amenorrhoea and galactorrhoea.

authors

- Iqbal A
- Novodvorsky P
- Lubina-Solomon A
- Kew FM
- Webster J

journal: Endocrinology, diabetes & metabolism case reports

journalAbbr: Endocrinol Diabetes Metab Case Rep

year: 2016

pubdate: 2016

volume: 2016

issue:

pages: 160006

abstract: UNLABELLED: Secondary amenorrhoea and galactorrhoea represent a common endocrine presentation. We report a case of an oestrogen-producing juvenile granulosa cell tumour (JGCT) of the ovary in a 16-year-old post-pubertal woman with hyperprolactinaemia amenorrhoea and galactorrhoea which resolved following surgical resection of the tumour. This patient presented with a 9-month history of secondary amenorrhoea and a 2-month history of galactorrhoea. Elevated serum prolactin at 7081 mIU/l and suppressed gonadotropins (LH <0.1 U/l; FSH <0.1 U/l) were detected. Serum oestradiol was significantly elevated at 7442 pmol/l with undetectable β-human chorionic gonadotropin. MRI showed a bulky pituitary with no visible adenoma. MRI of the abdomen showed a 4.8 cm mass arising from the right ovary with no evidence of metastatic disease. Serum inhibin B was elevated at 2735 ng/l. A right salpingo-oophorectomy was performed, and histology confirmed the diagnosis of a JGCT, stage International Federation of Gynaecology and Obstetrics 1A. Immunohistochemical staining for prolactin was negative. Post-operatively, oestrogen and prolactin levels

were normalised, and she subsequently had a successful pregnancy. In summary, we present a case of an oestrogen-secreting JGCT with hyperprolactinaemia manifesting clinically with galactorrhoea and secondary amenorrhoea. We postulate that observed hyperprolactinaemia was caused by oestrogenic stimulation of pituitary lactotroph cells, a biochemical state analogous to pregnancy. To the best of our knowledge, this is the first report of hyperprolactinaemia as a result of excessive oestrogen production in the context of a JGCT. LEARNING POINTS: Hyperprolactinaemia with bilateral galactorrhoea and secondary amenorrhoea has a wide differential diagnosis and is not always caused by a prolactin secreting pituitary adenoma.Significantly elevated serum oestradiol levels in the range seen in this case, in the absence of pregnancy, are indicative of an oestrogen-secreting tumour.JGCTs are rare hormonally active ovarian neoplasms mostly secreting steroid hormones.Serum inhibin can be used as a granulosa cell-specific tumour marker.JGCTs have an excellent prognosis in the early stages of the disease.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4815277/

Record 1153

pmcid: PMC4866215

pmid: 27190917

doi: 10.7860/JCDR/2016/16758.7640

title: Male Breast Abscess Secondary to Actinomycosis: A Case Report.

authors

- Mahendiran SA
- Leibman AJ
- Kornmehl AS

journal: Journal of clinical and diagnostic research : JCDR

journalAbbr: J Clin Diagn Res

year: 2016

pubdate: 2016 Apr

volume: 10

issue: 4

pages: TD05-TD07

abstract: Primary breast actinomycosis is a rare condition that has been previously reported in the female breast. Male breast infection is uncommon and most often associated with trauma to the skin or predisposing conditions like diabetes. We report the first case to our knowledge of primary breast actinomycosis in the male breast caused by Actinomycesneuii (A. neuii), a rare strain of Actinomyces. Mammography demonstrated periareolar skin thickening with a mottled pattern. Sonography showed multiple small cystic structures. Definitive diagnosis was made by culture of the nipple discharge.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4866215/

Record 1154

pmcid: PMC5080252

pmid: 27803541

doi: 10.1007/s13224-016-0864-4

title: McCune Albright Syndrome from Gynaecological Perspective.

authors

- Mishra A
- Purvar R

- Malik S
- Agarwal K
- Gera R
- Sridhar S

journal: Journal of obstetrics and gynaecology of India

journalAbbr: J Obstet Gynaecol India

year: 2016

pubdate: 2016 Oct

volume: 66

issue: Suppl 2

pages: 672-674

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5080252/

Record 1155

pmcid: PMC5108397

pmid: 27904534

doi: 10.5114/aoms.2016.62917

title: Metaplastic breast cancer with rapidly progressive recurrence in a young woman: case report and review of the literature.

authors

- Mituś JW
- Sas-Korczyńska B
- Kruczak A
- Jasiówka M
- Ryś J

journal: Archives of medical science : AMS

journalAbbr: Arch Med Sci

year: 2016

pubdate: 2016 Dec 1

volume: 12

issue: 6

pages: 1384-1388

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5108397/

Record 1156

pmcid: PMC6788628

pmid: 27268909

doi: 10.1016/j.tcb.2016.05.005

title: Microenvironmental Control of Adipocyte Fate and Function.

authors

- Pope BD
- Warren CR
- Parker KK
- Cowan CA

journal: Trends in cell biology

journalAbbr: Trends Cell Biol

year: 2016

pubdate: 2016 Oct

volume: 26

issue: 10

pages: 745-755

abstract: The properties of tissue-specific microenvironments vary widely in the human body and demonstrably influence the structure and function of many cell types. Adipocytes are no exception, responding to cues in specialized niches to perform vital metabolic and endocrine functions. The adipose microenvironment is remodeled during tissue expansion to maintain the structural and functional integrity of the tissue and disrupted remodeling in obesity contributes to the progression of metabolic syndrome, breast cancer, and other malignancies. The increasing incidence of these obesity-related diseases and the recent focus on improved in vitro models of human tissue biology underscore growing interest in the regulatory role of adipocyte microenvironments in health and disease.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6788628/

Record 1157

pmcid: PMC5110447

pmid: 26920904

doi: 10.1007/8904_2016_527

title: Multidisciplinary Team Approach Is Key for Managing Pregnancy and Delivery in Patient with Rare, Complex MPS I.

authors

- Troko J
- Poonawala Y
- Geberhiwot T
- Martin B

journal: JIMD reports

journalAbbr: JIMD Rep

year: 2016

pubdate: 2016

volume: 30

issue:

pages: 1-5

abstract: A 23-year-old primiparous lady (Ms S) was referred to preconception clinic with known Hurler-Scheie syndrome (mucopolysaccharidosis 1). Ms S had been under the care of the adult inherited metabolic disorder physicians prior to becoming pregnant. She and her partner received prenatal counselling and following

spontaneous conception was closely managed by a multidisciplinary team involving foetomaternal obstetricians, anaesthetists, cardiologists, geneticists and endocrinologists in two tertiary referral hospitals throughout her pregnancy. She went on to deliver a live male child at 37/40 by elective caesarean section. As far as we are aware, this is the first case report of a term pregnancy in a woman with moderate to severe mucopolysaccharidosis 1 (MPS 1).

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5110447/

Record 1158

pmcid: PMC4927014

pmid: 27408516

doi:

title: Pharmaceutical Approval Update.

authors

- Choy M

journal: P & T : a peer-reviewed journal for formulary management

journalAbbr: P T

year: 2016

pubdate: 2016 Jul

volume: 41

issue: 7

pages: 416-41

abstract: Defibrotide sodium (Defitelio) for hepatic veno-occlusive disease; emtricitabine/tenofovir alafenamide (Descovy) for human immunodeficiency virus infection; and infliximabdyyb (Inflectra) for Crohn's disease, ulcerative colitis, rheumatoid arthritis, ankylosing spondylitis, psoriatic arthritis, and plaque psoriasis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4927014/

Record 1159

pmcid: PMC8265874

pmid: 26836292

doi: 10.1002/14651858.CD011522.pub2

title: Pregabalin for decreasing pancreatic pain in chronic pancreatitis.

authors

- Gurusamy KS
- Lusuku C
- Davidson BR

journal: The Cochrane database of systematic reviews

journalAbbr: Cochrane Database Syst Rev

year: 2016

pubdate: 2016 Feb 2

volume: 2

issue: 2

pages: CD011522

abstract: BACKGROUND: Chronic abdominal pain is one of the major symptoms in people with chronic pancreatitis. The role of pregabalin in people with chronic pancreatic pain due to chronic pancreatitis is uncertain. OBJECTIVES: To assess the benefits and harms of pregabalin in people with chronic abdominal pain due to chronic pancreatitis. SEARCH METHODS: We searched the Cochrane Central Register of Controlled Trials (CENTRAL) in The Cochrane Library 2015, issue 6, and MEDLINE, EMBASE, Science Citation Index Expanded, trials registers until June 2015. We also searched the references of included trials to identify further trials. SELECTION CRITERIA: We considered only randomised controlled trials (RCT) performed in people with chronic pancreatic pain due to chronic pancreatitis, irrespective of language, blinding, or publication status for inclusion in the review. DATA COLLECTION AND ANALYSIS: Two review authors independently identified trials and independently extracted data. We calculated the risk ratio (RR) or mean difference (MD) with 95% confidence intervals (CI) with RevMan 5, based on intention-to-treat analysis. MAIN RESULTS: Only one study, funded by Pfizer, met the inclusion criteria for the review. A total of 64 participants (with chronic pain due to chronic pancreatitis) were randomly assigned to receive escalating doses of pregabalin (150 mg per day to 600 mg per day; 34 participants) or matching placebo (30 participants). Participants received pregabalin or placebo for three weeks on an outpatient basis; the outcomes were measured at the end of the treatment (i.e. three weeks from commencement of treatment). Potential participants taking concomitant analgesic medication and expected to stay on a stable regime during the trial were allowed to enter the study. This trial was at low risk of bias. The overall quality of evidence was low or moderate.Only the short-term outcomes were available in this trial. The medium and long-term outcomes, number of work days lost, and length of hospital stay due to admissions for pain control were not available. This trial found that the changes in opiate use (MD -26.00 mg; 95% CI -47.36 to -4.64; participants = 64; moderate-quality evidence), and pain score percentage changes from baseline (MD -12.00; 95% CI -21.82 to -2.18; participants = 64; moderate-quality evidence) were better in participants taking pregabalin compared to those taking placebo. This trial also found that there were more adverse events in participants taking pregabalin compared to those taking placebo (RR 1.71; 95% CI 1.20 to 2.43; participants = 64). The differences between pregabalin and placebo were imprecise for short-term health-related quality of life measured with the EORTC CLQ-30 questionnaire (MD 11.40; 95% CI -3.28 to 26.08; participants = 64; moderate-quality evidence), proportion of people with serious adverse events (RR 1.76; 95% CI 0.35 to 8.96; participants = 64; low-quality evidence), and proportion of people requiring hospital admissions (RR 0.44; 95% CI 0.04 to 4.62; participants = 64; low quality evidence). AUTHORS' CONCLUSIONS: Based on low- to moderate-quality evidence, short-term use of pregabalin decreases short-term opiate use, and short-term pain scores, but increases the adverse events compared to placebo, in people with chronic pain due to chronic pancreatitis. The clinical implication of the decreases in short-term opiate use and short-term pain scores is not known.Future trials assessing the role of pregabalin in decreasing chronic pain in chronic pancreatitis should assess the medium- or long-term effects of pregabalin and should include outcomes such as, quality of life, treatment-related adverse events, number of work days lost, number of hospital admissions, and the length of hospital stay, in addition to pain scores, to assess the clinical and socioeconomic impact.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8265874/

Record 1160

pmcid: PMC4763798

pmid: 26893404

doi: 10.12659/ajcr.896264

title: Primary Breast Lymphoma in a Woman: A Case Report and Review of the Literature.

authors

- Jabbour G
- El-Mabrok G
- Al-Thani H
- El-Menyar A
- Al Hijji I
- Napaki S

journal: The American journal of case reports

journalAbbr: Am J Case Rep

year: 2016

pubdate: 2016 Feb 19

volume: 17

issue:

pages: 97-103

abstract: BACKGROUND: Primary breast lymphoma (PBL) is an unusual clinical entity accounting for 0.4-0.5% of all breast neoplasms. The usual presentation includes a painless palpable mass similar to that of breast carcinoma. Diffuse large B-cell lymphoma (DLBCL) is the most common identifiable type of PBL based on the histopathological examination. CASE REPORT: We report an unusual case of a 43-year-old Indonesian woman who presented with a 7-month history of a painless mass in the left breast. A core needle biopsy revealed diffuse infiltration of large atypical lymphoid cells. The immuno-histochemical biomarkers confirmed the diagnosis of a DLBCL. A bone scan showed no evidence of bone metastasis. It was treated non-surgically, based on the decision of the breast multidisciplinary team (MDT). The patient was treated with 4 cycles of combination chemotherapy with R-CODOX/IVAC. A follow-up PET scan revealed non-significant mild F-18 fluorodeoxyglucose (FDG) uptake at the periphery of the residual left breast mass, indicating a radiologically favorable response. CONCLUSIONS: Early and accurate diagnosis of PBL is crucial for selecting the appropriate MDT treatment strategies to avert potentially harmful surgical interventions.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4763798/

Record 1161

pmcid: PMC11376434

pmid: 26410174

doi: 10.1016/j.contraception.2015.09.010

title: Progestogen-only contraceptive use among breastfeeding women: a systematic review.

authors

- Phillips SJ
- Tepper NK
- Kapp N
- Nanda K
- Temmerman M
- Curtis KM

journal: Contraception

journalAbbr: Contraception

year: 2016

pubdate: 2016 Sep

volume: 94

issue: 3

pages: 226-52

abstract: BACKGROUND: Postpartum women need effective contraception. Concerns have been raised that use of progestogen-only contraceptives (POCs) may affect breastfeeding performance and infant health outcomes. OBJECTIVES: We investigated the clinical outcomes of breastfeeding duration, initiation of supplemental feeding and weaning, as well as infant outcomes including infant growth, health and development among breastfeeding women using POCs compared with breastfeeding women not using POCs. SEARCH STRATEGY: We searched the PubMed database for all articles published from database inception through

December 2014. SELECTION CRITERIA: We included primary research studies of breastfeeding women of any age or parity who received POCs, including progestogen-only pills, injectables, implants or hormonal intrauterine devices (IUDs). The main outcomes were breastfeeding performance (as measured by initiation, continuation, frequency and exclusivity of breastfeeding) and infant health (as measured by growth, development or adverse health effects). RESULTS: Forty-nine articles reporting on 47 different studies were identified that investigated the use of POCs in breastfeeding women and reported clinically relevant outcomes of infant growth, health or breastfeeding performance. Studies ranged from poor to fair methodological quality and generally failed to show negative effects of the use of POCs on breastfeeding outcomes or on infant growth or development. One randomized controlled trial (RCT) raises concerns that immediate insertion of the levonorgestrel IUD postpartum may be associated with poorer breastfeeding performance when compared with delayed insertion, although two other RCTs evaluating early etonogestrel implants compared with delayed initiation of implants or depot medroxyprogesterone acetate failed to find such an association. CONCLUSION: The preponderance of evidence fails to demonstrate adverse breastfeeding outcomes or negative health outcomes in infants such as restricted growth, health problems or impaired development. Evidence newly added to this review was largely consistent with previous evidence.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC11376434/

Record 1162

pmcid: PMC4807181

pmid: 27019818

doi: 10.5999/aps.2016.43.2.218

title: Pseudoangiomatous Stromal Hyperplasia Presenting as Rapidly Growing Bilateral Breast Enlargement Refractory to Surgical Excision.

authors

- Lee JW
- Jung GS
- Kim JB
- Choi KY
- Chung HY
- Cho BC
- Park JY
- Kim HJ
- Park HY
- Yang JD

journal: Archives of plastic surgery

journalAbbr: Arch Plast Surg

year: 2016

pubdate: 2016 Mar

volume: 43

issue: 2

pages: 218-21

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4807181/

Record 1163

pmcid: PMC6705542

pmid: 27413150

doi: 10.1523/JNEUROSCI.4061-15.2016

title: Serotonin and Antidepressant SSRIs Inhibit Rat Neuroendocrine Dopamine Neurons: Parallel Actions in the Lactotrophic Axis.

authors

- Lyons DJ
- Ammari R
- Hellysaz A
- Broberger C

journal: The Journal of neuroscience : the official journal of the Society for Neuroscience

journalAbbr: J Neurosci

year: 2016

pubdate: 2016 Jul 13

volume: 36

issue: 28

pages: 7392-406

abstract: UNLABELLED: Selective serotonin reuptake inhibitors (SSRIs) are commonly prescribed for depression, but sexual side effects often compromise compliance. These reproductive dysfunctions are likely mediated by elevations of the hormone prolactin. Yet, how serotonin (5-HT) and SSRIs cause changes in prolactin secretion is not known. Here, using in vitro whole-cell patch-clamp recordings, we show that 5-HT hyperpolarizes and abolishes phasic discharge in rat neuroendocrine tuberoinfundibular dopamine (TIDA) neurons, the main inhibitor of prolactin secretion. This process is underpinned by 5-HT1A receptor-mediated activation of G-protein-coupled inwardly rectifying K(+)-like currents. We further demonstrate that the SSRIs, fluoxetine and sertraline, directly suppress TIDA neuron activity through parallel effects, independent of 5-HT transmission. This inhibition involves decreased intrinsic excitability and a slowing of TIDA network rhythms. These findings indicate that SSRIs may inhibit neuroendocrine dopamine release through both 5-HT-dependent and -independent actions, providing a mechanistic explanation for, and potential molecular targets for the amelioration of, the hyperprolactinemia and sexual dysfunction associated with these drugs. SIGNIFICANCE STATEMENT: Depression affects approximately one-tenth of the population and is commonly treated with selective serotonin reuptake inhibitors (SSRIs; e.g., Prozac). Yet, many patients withdraw from SSRI therapy due to sexual side effects (e.g., infertility, menstrual disturbances, and impotence). Although it is generally accepted that sexual side effects are due to the ability of these drugs to elevate blood levels of the hormone prolactin, the mechanism for this hormonal imbalance is not known. Here, we show that SSRIs can inhibit hypothalamic dopamine neurons that normally suppress the secretion of prolactin. Intriguingly this inhibition can be explained both by increased serotonin activity and also by parallel serotonin-independent actions.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6705542/

Record 1164

pmcid: PMC4805039

pmid: 26761949

doi: 10.4274/jcrpe.2764

title: Sex Hormone-Binding Globulin in Children and Adolescents.

authors

- Aydın B
- Winters SJ

journal: Journal of clinical research in pediatric endocrinology

journalAbbr: J Clin Res Pediatr Endocrinol

year: 2016

pubdate: 2016 Mar 5

volume: 8

issue: 1

pages: 1-12

abstract: Sex hormone-binding globulin (SHBG) is a circulating glycoprotein that transports testosterone and other steroids in the blood. Interest in SHBG has escalated in recent years because of its inverse association with obesity and insulin resistance, and because many studies have linked lower circulating levels of SHBG to metabolic syndrome, type 2 diabetes, nonalcoholic fatty liver disease, polycystic ovary syndrome, and early puberty. The purpose of this review is to summarize molecular, clinical, endocrine, and epidemiological findings to illustrate how measurement of plasma SHBG may be useful in clinical medicine in children.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4805039/

Record 1165

pmcid: PMC5094812

pmid: 26547319

doi: 10.1007/s12031-015-0674-7

title: Spinal and Bulbar Muscular Atrophy Overview.

authors

- Fischbeck KH

journal: Journal of molecular neuroscience : MN

journalAbbr: J Mol Neurosci

year: 2016

pubdate: 2016 Mar

volume: 58

issue: 3

pages: 317-20

abstract: Spinal and bulbar muscular atrophy is an X-linked neuromuscular disease caused by an expanded repeat in the androgen receptor gene. The mutant protein is toxic to motor neurons and muscle. The toxicity is ligand-dependent and likely involves aberrant interaction of the mutant androgen receptor with other nuclear factors leading to transcriptional dysregulation. Various therapeutic strategies have been effective in transgenic animal models, and the challenge now is to translate these strategies into safe and effective treatment in patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5094812/

Record 1166

pmcid: PMC5351485

pmid: 28331751

doi: 10.5152/tjbh.2016.3115

title: Subcutaneous Oleomas Following Sunflower Oil Injection: A Novel Case and Review of Literature.

authors

- Sarıca Ö
- Kayhan A
- Demirkürek HC
- İğdem AA

journal: The journal of breast health

journalAbbr: J Breast Health

year: 2016

pubdate: 2016 Jul

volume: 12

issue: 3

pages: 141-144

abstract: Liquid foreign material injection has been used as an early medical intervention since the end of nineteenth century for the augmentation of body shape. Nowadays, these types of procedures have been abandoned by health professionals due to late onset of serious complications. However, it is still misused by some subcultures such as bodybuilders, passive homosexuals, transsexuals, and patients with mental illness. This article discusses a male patient who injected himself with a large amount of sunflower oil, which became complicated by an inflammatory response-abscess formation and sclerosing lipogranuloma of breasts. The radiologic and pathologic signs of this entity are discussed with a review of the relevant literature. Lack of suspicion of this entity may cause a great delay in establishment of definitive diagnosis, giving rise to prominent morbidity and mortality. It is necessary to know the diagnosis and treatment of this phenomenon because illegal substances that cause factitial panniculitis are widely available on websites and threaten thousands of people, which is anecdotally referred in medical literature. Chronic or recurrent lesions of a bizarre or atypical morphology should alert the physician to this artificial phenomenon. Radiologic findings are most important criteria for diagnosis because self-injection is denied by the patient.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5351485/

Record 1167

pmcid: PMC5161216

pmid: 27938530

doi: 10.11138/gchir/2016.37.4.150

title: Surgical management of gynecomastia: experience of a general surgery center.

authors

- Longheu A
- Medas F
- Corrias F
- Farris S
- Tatti A
- Pisano G
- Erdas E
- Calò PG

journal: Il Giornale di chirurgia

journalAbbr: G Chir

year: 2016

pubdate: 2016 Jul-Aug

volume: 37

issue: 4

pages: 150-154

abstract: AIM: Gynecomastia is a common finding in male population of all ages. The aim of our study was to present our experience and goals in surgical treatment of gynecomastia. PATIENTS AND METHODS: Clinical records of patients affected by gynecomastia referred to our Department of Surgery between September 2008 and January 2015 were analyzed. 50 patients were included in this study. RESULTS: Gynecomastia was monolateral in 12 patients (24%) and bilateral in 38 (76%); idiopathic in 41 patients (82%) and secondary in 9 (18%). 39 patients (78%) underwent surgical operation under general anaesthesia, 11 (22%) under local anaesthesia. 3 patients (6%) presented recurrent disease. Webster technique was performed in 28 patients (56%), Davidson technique in 16 patients (32%); in 2 patients (4%) Pitanguy technique was performed and in 4 patients (8%) a mixed surgical technique was performed. Mean surgical time was 80.72±35.14 minutes, median postoperative stay was 1.46±0.88 days. 2 patients (4%) operated using Davidson technique developed a hematoma, 1 patient (2%) operated with the same technique developed hypertrophic scar. CONCLUSIONS: Several surgical techniques are described for surgical correction of gynecomastia. If performed by skilled general surgeons surgical treatment of gynecomastia is safe and permits to reach satisfactory aesthetic results.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5161216/

Record 1168

pmcid: PMC4915214

pmid: 27331059

doi: 10.17795/nmsjournal32990

title: The Effect of Spiritual Intervention on Postmenopausal Depression in Women Referred to Urban Healthcare Centers in Isfahan: A Double-Blind Clinical Trial.

authors

- Shafiee Z
- Zandiyeh Z
- Moeini M
- Gholami A

journal: Nursing and midwifery studies

journalAbbr: Nurs Midwifery Stud

year: 2016

pubdate: 2016 Mar

volume: 5

issue: 1

pages: e32990

abstract: BACKGROUND: Depression is not only common after menopause, but also affects postmenopausal women more than other women. Some studies show the positive effects of spiritual intervention on postmenopausal women and depressed patients. However, there is inadequate experimental data for supporting the effectiveness of such interventions. OBJECTIVES: This study investigated the effect of a spiritual intervention on postmenopausal depression in women referred to urban healthcare centers in Isfahan, Iran. PATIENTS AND METHODS: A randomized controlled clinical trial was conducted on postmenopausal women referred to the healthcare centers of Isfahan. Sixty-four women with postmenopausal depression were assigned randomly into an experimental group (n = 32) and a control group (n = 32). The experimental group received eight sessions of spiritual intervention while the control group received two sessions of training on healthy diet for postmenopausal women. All subjects in the experimental group and the control group responded to the Beck's depression inventory at the start of the study, at the end of the fourth week, and a month after the last

educational session. In addition to descriptive statistics, the chi-square test, independent samples t-test and repeated measures analysis of variance were used to analyze the data. RESULTS: Before the intervention, the study groups did not differ significantly in terms of mean depression scores (20.76 ± 4.61 vs. 19.58 ± 5.27, P = 0.33). However, immediately after intervention and after one month, the mean depression scores of 11.01 ± 7.85 and 11.21 ± 9.23 in the experimental group were significantly lower than the control group (19.22 ± 4.94 and 19.34 ± 4.92, respectively) (P = 0.001). In repeated measures analysis of variance, Mauchly's test of sphericity was not significant (P = 0.672), and in the test of within-subjects effects, a significant interaction was found between the spiritual intervention and time. CONCLUSIONS: Spiritual intervention effectively could reduce the severity of postmenopausal depression. Considering the high prevalence of depression in postmenopausal women and the effectiveness, simplicity, and affordability of spiritual intervention, using such interventions in postmenopausal women is recommended.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4915214/

Record 1169

pmcid: PMC4743592

pmid: 26595752

doi: 10.3109/15622975.2015.1085598

title: The World Federation of Societies of Biological Psychiatry (WFSBP) guidelines for the treatment of adolescent sexual offenders with paraphilic disorders.

authors

- Thibaut F
- Bradford JM
- Briken P
- De La Barra F
- Häßler F
- Cosyns P

journal: The world journal of biological psychiatry : the official journal of the World Federation of Societies of Biological Psychiatry

journalAbbr: World J Biol Psychiatry

year: 2016

pubdate: 2016

volume: 17

issue: 1

pages: 2-38

abstract: The primary aim of these guidelines was to evaluate the role of pharmacological agents in the treatment of adolescents with paraphilic disorders who are also sexual offenders or at-risk of sexual offending. Psychotherapeutic and psychosocial treatments were also reviewed. Adolescents with paraphilic disorders specifically present a different therapeutic challenge as compared to adults. In part, the challenge relates to adolescents being in various stages of puberty and development, which may limit the use of certain pharmacological agents due to their potential side effects. In addition, most of the published treatment programmes have used cognitive behavioural interventions, family therapies and psychoeducational interventions. Psychological treatment is predicated in adolescents on the notion that sexually deviant behaviour can be controlled by the offender, and that more adaptive behaviours can be learned. The main purposes of these guidelines are to improve the quality of care and to aid physicians in their clinical decisions. These guidelines brought together different expert views and involved an extensive literature research. Each treatment recommendation was evaluated and discussed with respect to the strength of evidence for efficacy, safety, tolerability and feasibility. An algorithm is proposed for the treatment of paraphilic disorders in adolescent sexual offenders or those who are at risk.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4743592/

Record 1170

pmcid: PMC5345994

pmid: 26680582

doi: 10.1159/000438895

title: Treatment of Peripheral Precocious Puberty.

authors

- Schoelwer M
- Eugster EA

journal: Endocrine development

journalAbbr: Endocr Dev

year: 2016

pubdate: 2016

volume: 29

issue:

pages: 230-9

abstract: There are many etiologies of peripheral precocious puberty (PPP) with diverse manifestations resulting from exposure to androgens, estrogens, or both. The clinical presentation depends on the underlying process and may be acute or gradual. The primary goals of therapy are to halt pubertal development and restore sex steroids to prepubertal values. Attenuation of linear growth velocity and rate of skeletal maturation in order to maximize height potential are additional considerations for many patients. McCune-Albright syndrome (MAS) and familial male-limited precocious puberty (FMPP) represent rare causes of PPP that arise from activating mutations in GNAS1 and the LH receptor gene, respectively. Several different therapeutic approaches have been investigated for both conditions with variable success. Experience to date suggests that the ideal therapy for precocious puberty secondary to MAS in girls remains elusive. In contrast, while the number of treated patients remains small, several successful therapeutic options for FMPP are available.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5345994/

Record 1171

pmcid: PMC4756524

pmid: 26884053

doi: 10.1186/s12903-016-0182-3

title: What are patients' expectations of orthodontic treatment: a systematic review.

authors

- Yao J
- Li DD
- Yang YQ
- McGrath CP
- Mattheos N

journal: BMC oral health

journalAbbr: BMC Oral Health

year: 2016

pubdate: 2016 Feb 17

volume: 16

issue:

pages: 19

abstract: BACKGROUND: What patients expect to happen during treatment or benefit from the treatment might influence the subsequent factors such as treatment outcome, patient satisfaction, patient's cooperation as well as compliance. The aim of this systematic review is to assess the role of patients' expectations from orthodontic treatment. METHODS: A systematic literature search of four databases Pubmed, Cochrane, Web of Science and PsychINFO was conducted following PRISMA guidelines. Studies reporting expectations regarding orthodontic treatment were selected and a narrative review was conducted. The quality of study was rated according to STROBE statements and the methodology as well as key findings were summarized. RESULTS: Thirteen studies (14 papers) were finally included for analysis. Among them, only one was a randomized control trial, while the rest included one cohort study, two questionnaire-developments and ten cross-sectional studies. The STROBE quality of reporting scores of the studies ranged from 12 to 18. Seven papers described expectations of the treatment experiences, along with seven talking about benefit expectations from the treatment. Dental appearance and function improvement were most expected in studies relate to the treatment benefits. CONCLUSIONS: Orthodontics appears to have adopted various standardized questionnaires. However, most of them are poor in the quality of methodology and results analyses, which prohibit synthesizing sufficient evidence to help identify which factors influence patient expectations. The evidence of "expectations" affecting treatment outcomes is not found in current research. Future studies are needed to better understand the impact of "expectation" on the treatment both theoretically and experimentally.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4756524/

Record 1172

pmcid: PMC5109383

pmid: 27853685

doi:

title: What's Preventing Us to Get More Attraction: The Fear of Aesthetic Surgery.

authors

- Leitermann M
- Hoffmann K
- Kasten E

journal: World journal of plastic surgery

journalAbbr: World J Plast Surg

year: 2016

pubdate: 2016 Sep

volume: 5

issue: 3

pages: 226-235

abstract: BACKGROUND: Nowadays, with the help of cosmetic surgery almost every woman and man can achieve a highly attractive appearance. The question is, why so many people do not take advantage of these opportunities? This pilot-study investigates individual attitudes of people towards aesthetic plastic surgery. METHODS: A questionnaire was developed which combined self-developed items for a measurement of attitudes towards plastic surgery. In addition, items of the "Freiburger Personality Inventory" (FPI-R) were used. The study was conducted in Hamburg/Germany. 104One hundred and four test persons participated in the survey (81 females, 23 males, age 20-30 years). Eighty six of the participants (82.7%) had an A-level as degree of education, 14.4% achieved the secondary school certificate and 2.9% had completed their bachelor on a high

school. RESULTS: The data supported the hypothesis that people who are unsatisfied with their body appearance showed more willingness for a surgical intervention. On the other hand, fear of complications and pain as far as anxiety before an unsatisfactory result hinders them from a decision for an intervention. Significant correlations with regard to extraversion-introversion and the education level were not found. Females showed more willingness regarding an intervention than men. Gender-specific differences concerning the cost factor were not found. CONCLUSION: Interestingly more than 65% of the total sample felt dissatisfaction with a specific body part and are thus target of aesthetic surgery. The yellow press often reports about failed cosmetic surgery, especially in VIP-persons. Aesthetic surgery should keep working to reduce unwarranted fears of people toward these kinds of operations.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5109383/

Record 1173

pmcid: PMC4643459

pmid: 26433926

doi: 10.1016/j.ijscr.2015.09.026

title: A pre-pubertal girl with giant juvenile fibroadenoma: A rare case report.

authors

- Gaurav K
- Chandra G
- Neelam K
- Kumar S
- Singla H
- Yadav SK

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2015

pubdate: 2015

volume: 16

issue:

pages: 87-9

abstract: INTRODUCTION: Fibroadenomas are benign neoplasms usually arising between the ages of 15-25 years. Approximately 0.4% fibroadenomas arise in juvenile age group. Usually the diagnosis is straightforward by clinical examination and FNAC. But sometimes rapid growth and giant size may pose difficulty in clinical approach. CASE PRESENTATION: In this paper we are presenting a rare case of giant juvenile fibroadenoma in a 10 years old girl which was diagnosed by FNAC and treated by excision. Diagnosis was confirmed by histopathology. DISCUSSION: Giant juvenile fibroadenomas are over 5cm in diameter and tend to show rapid growth mimicking a carcinoma. But histological features are similar to smaller fibroadenomas and can be enucleated. CONCLUSION: Through this case we want to emphasize that these giant benign neoplasms should be suspected in any pre-pubertal girl with breast lump and should always be treated with breast conserving surgery.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4643459/

Record 1174

pmcid: PMC4561109

pmid: 26366315

doi: 10.1155/2015/439239

title: A Rare Cause of Prepubertal Gynecomastia: Sertoli Cell Tumor.

authors

- Dursun F
- Su Dur ŞM
- Şahin C
- Kırmızıbekmez H
- Karabulut MH
- Yörük A

journal: Case reports in pediatrics

journalAbbr: Case Rep Pediatr

year: 2015

pubdate: 2015

volume: 2015

issue:

pages: 439239

abstract: Prepubertal gynecomastia due to testis tumors is a very rare condition. Nearly 5% of the patients with testicular mass present with gynecomastia. Sertoli cell tumors are sporadic in 60% of the reported cases, while the remaining is a component of multiple neoplasia syndromes such as Peutz-Jeghers syndrome and Carney complex. We present a 4-year-old boy with gynecomastia due to Sertoli cell tumor with no evidence of Peutz-Jeghers syndrome or Carney complex.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4561109/

Record 1175

pmcid: PMC4727688

pmid: 26894004

doi: 10.1097/GOX.0000000000000561

title: A Subglandular Breast Cerebrospinal Fluid Pseudocyst Following Postsurgical Shunt Migration.

authors

- Mlynek K
- Frautschi R
- Halasa B
- Kwiecien G
- Papay F

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2015

pubdate: 2015 Dec

volume: 3

issue: 12

pages: e579

abstract: Cerebrospinal fluid (CSF) drainage catheters have been associated with numerous complications in various anatomic locations, because of migration, infection, and obstruction. However, breast-related CSF shunt

complications tend to occur infrequently or have seldom been reported in the empirical literature. Therefore, a case is presented detailing a breast pseudocyst caused by migration and subsequent coiling of a ventriculoperitoneal shunt in the right breast pocket. To the best of the authors' knowledge, this is the first case that has been reported in the peer-reviewed literature of a pseudocyst resulting from a CSF drainage catheter coiling around the breast implant post pancreaticoduodenectomy. Moreover, this case highlights the importance of cross-disciplinary procedural awareness, particularly in regards to breast, ventriculoperitoneal shunt, and pancreatic procedures.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4727688/

Record 1176

pmcid: PMC4331402

pmid: 25713759

doi: 10.1186/s40064-015-0808-y

title: A systematic review of non-hormonal treatments of vasomotor symptoms in climacteric and cancer patients.

authors

- Drewe J
- Bucher KA
- Zahner C

journal: SpringerPlus

journalAbbr: Springerplus

year: 2015

pubdate: 2015

volume: 4

issue:

pages: 65

abstract: The cardinal climacteric symptoms of hot flushes and night sweats affect 24-93% of all women during the physiological transition from reproductive to post-reproductive life. Though efficacious, hormonal therapy and partial oestrogenic compounds are linked to a significant increase in breast cancer. Non-hormonal treatments are thus greatly appreciated. This systematic review of published hormonal and non-hormonal treatments for climacteric, and breast and prostate cancer-associated hot flushes, examines clinical efficacy and therapy-related cancer risk modulation. A PubMed search included literature up to June 19, 2014 without limits for initial dates or language, with the search terms, (hot flush* OR hot flash*) AND (clinical trial* OR clinical stud*) AND (randomi* OR observational) NOT review). Retrieved references identified further papers. The focus was on hot flushes; other symptoms (night sweats, irritability, etc.) were not specifically screened. Included were some 610 clinical studies where a measured effect of the intervention, intensity and severity were documented, and where patients received treatment of pharmaceutical quality. Only 147 of these references described studies with alternative non-hormonal treatments in post-menopausal women and in breast and prostate cancer survivors; these results are presented in Additional file 1. The most effective hot flush treatment is oestrogenic hormones, or a combination of oestrogen and progestins, though benefits are partially outweighed by a significantly increased risk for breast cancer development. This review illustrates that certain non-hormonal treatments, including selective serotonin reuptake inhibitors, gabapentin/pregabalin, and Cimicifuga racemosa extracts, show a positive risk-benefit ratio. Key pointsSeveral non-hormonal alternatives to hormonal therapy have been established and registered for the treatment of vasomotor climacteric symptoms in peri- and post-menopausal women.There are indications that non-hormonal treatments are useful alternatives in patients with a history of breast and prostate cancer. However, confirmation by larger clinical trials is required.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4331402/

Record 1177

pmcid: PMC4405523

pmid: 25869016

doi: 10.1007/s11606-015-3271-0

title: Abstracts from the 38th annual meeting of the society of general internal medicine.

authors

journal: Journal of general internal medicine

journalAbbr: J Gen Intern Med

year: 2015

pubdate: 2015 Apr

volume: 30 Suppl 2

issue: Suppl 2

pages: 45-551

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4405523/

Record 1178

pmcid: PMC4347046

pmid: 25741497

doi: 10.4174/astr.2015.88.3.166

title: An idiopathic gigantomastia.

authors

- Cho MJ
- Yang JH
- Choi HG
- Kim WS
- Yu YB
- Park KS

journal: Annals of surgical treatment and research

journalAbbr: Ann Surg Treat Res

year: 2015

pubdate: 2015 Mar

volume: 88

issue: 3

pages: 166-9

abstract: Gigantomastia is a rare condition characterized by excessive breast growth. It has been reported that the majority of gigantomastia cases occur during either pregnancy or puberty. We were presented with a rare case of gigantomastia associated with neither pregnancy nor puberty, and successfully treated it with reduction mammaplasty and free nipple graft. This idiopathic gigantomastia is the very first case in Korea, and adds to the worldwide total of 9 reported cases.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4347046/

Record 1179

pmcid: PMC4372884

pmid: 25814815

doi: 10.4103/0970-2113.152665

title: An unusual case of mesothelioma.

authors

- Gothi D
- Verma A
- Sah RB

journal: Lung India : official organ of Indian Chest Society

journalAbbr: Lung India

year: 2015

pubdate: 2015 Mar-Apr

volume: 32

issue: 2

pages: 194-5

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4372884/

Record 1180

pmcid: PMC4359241

pmid: 25595907

doi: 10.18632/oncotarget.2831

title: Androgen receptors beyond prostate cancer: an old marker as a new target.

authors

- Munoz J
- Wheler JJ
- Kurzrock R

journal: Oncotarget

journalAbbr: Oncotarget

year: 2015

pubdate: 2015 Jan 20

volume: 6

issue: 2

pages: 592-603

abstract: Androgen receptors (ARs) play a critical role in the development of prostate cancer. Targeting ARs results in important salutary effects in this malignancy. Despite mounting evidence that ARs also participate in the pathogenesis and/or progression of diverse tumors, exploring the impact of hormonal manipulation of these receptors has not been widely pursued beyond prostate cancer. This review describes patterns of AR expression in a spectrum of cancers, and the potential to exploit this knowledge in the clinical therapeutic setting.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4359241/

Record 1181

pmcid: PMC4147033

pmid: 24582702

doi: 10.1016/j.jvs.2014.01.032

title: Aromatase is required for female abdominal aortic aneurysm protection.

authors

- Johnston WF
- Salmon M
- Su G
- Lu G
- Ailawadi G
- Upchurch GR Jr

journal: Journal of vascular surgery

journalAbbr: J Vasc Surg

year: 2015

pubdate: 2015 Jun

volume: 61

issue: 6

pages: 1565-74.e1-4

abstract: OBJECTIVE: The protective effects of female gender on the development of abdominal aortic aneurysms (AAAs) have been attributed to anti-inflammatory effects of estrogen. Estrogen synthesis is dependent on the enzyme aromatase, which is located both centrally in the ovaries and peripherally in adipose tissue, bone, and vascular smooth muscle cells. It is hypothesized that deletion of aromatase in both ovarian and peripheral tissues would diminish the protective effect of female gender and would be associated with increased aortic diameter in female mice. METHODS: Male and female 8- to 10-week-old mice with aromatase (wild type: WT) and without aromatase (ArKO) underwent elastase aortic perfusion with aortic harvest 14 days following. For the contribution of central and peripheral estrogen conversion to be evaluated, female WT mice were compared with female WT and ArKO mice that had undergone ovariectomy (ovx) at 6 weeks followed by elastase perfusion at 8 to 10 weeks. At aortic harvest, maximal aortic dilation was measured and samples were collected for immunohistochemistry and protein analysis. Serum was collected for serum estradiol concentrations. Groups were compared with analysis of variance. Human and mouse AAA cross sections were analyzed with confocal immunohistochemistry for aromatase, smooth muscle markers, and macrophage markers. RESULTS: Female WT mice had significant reduction in aortic dilation compared with male WT mice (F WT, 51.5% ± 15.1% vs M WT, 78.7% ± 14.9%; P < .005). The protective effects of female gender were completely eliminated with deletion of aromatase (F ArKO, 82.6% ± 13.8%; P < .05 vs F WT). Ovariectomy increased aortic dilation in WT mice (F WT ovx, 70.6% ± 11.7%; P < .05 vs F WT). Aromatase deletion with ovariectomy further increased aortic dilation compared with WT ovx mice (F ArKO ovx, 87.3% ± 14.7%, P < .001 vs F WT and P < .05 vs F WT ovx). Accordingly, female ArKO ovx mice had significantly higher levels of the proinflammatory cytokines monocyte chemoattractant protein 1 and interleukin-1β and were associated with increased macrophage staining and decreased elastin staining. Regarding serum hormone levels, decreasing estradiol levels correlated with increasing aortic diameter (R = -0.565; P < .01). By confocal immunohistochemistry, both human and mouse AAA smooth muscle cells (smooth muscle α-actin positive) and macrophages (CD68 positive or Mac-2 positive) expressed aromatase. CONCLUSIONS: The protective effect of female gender on AAAs is due to estrogen synthesis and requires the presence of both ovarian and extragonadal/peripheral aromatase. Peripheral estrogen synthesis accounts for roughly half of the protective effect of female gender. If peripheral aromatase could be targeted, high levels of local estrogen could be

produced and may avoid the side effects of systemic estrogen.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4147033/

Record 1182

pmcid: PMC4678918

pmid: 26670406

doi: 10.1016/j.bjhh.2015.08.007

title: Axillary high-grade B-cell non-Hodgkin lymphoma presenting under the guise of inflammatory breast carcinoma.

authors

- Ribas C
- Navarro MA
- Z B Pope L
- Hornburg G

journal: Revista brasileira de hematologia e hemoterapia

journalAbbr: Rev Bras Hematol Hemoter

year: 2015

pubdate: 2015 Nov-Dec

volume: 37

issue: 6

pages: 417-9

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4678918/

Record 1183

pmcid: PMC4403929

pmid: 25888057

doi: 10.1186/s13058-015-0565-4

title: Background risk of breast cancer and the association between physical activity and mammographic density.

authors

- Trinh T
- Eriksson M
- Darabi H
- Bonn SE
- Brand JS
- Cuzick J
- Czene K
- Sjölander A
- Bälter K
- Hall P

journal: Breast cancer research : BCR

journalAbbr: Breast Cancer Res

year: 2015

pubdate: 2015 Apr 2

volume: 17

issue: 1

pages: 50

abstract: INTRODUCTION: High physical activity has been shown to decrease the risk of breast cancer, potentially by a mechanism that also reduces mammographic density. We tested the hypothesis that the risk of developing breast cancer in the next 10 years according to the Tyrer-Cuzick prediction model influences the association between physical activity and mammographic density. METHODS: We conducted a population-based cross-sectional study of 38,913 Swedish women aged 40-74 years. Physical activity was assessed using the validated web-questionnaire Active-Q and mammographic density was measured by the fully automated volumetric Volpara method. The 10-year risk of breast cancer was estimated using the Tyrer-Cuzick (TC) prediction model. Linear regression analyses were performed to assess the association between physical activity and volumetric mammographic density and the potential interaction with the TC breast cancer risk. RESULTS: Overall, high physical activity was associated with lower absolute dense volume. As compared to women with the lowest total activity level (<40 metabolic equivalent hours [MET-h] per day), women with the highest total activity level (≥50 MET-h/day) had an estimated 3.4 cm(3) (95% confidence interval, 2.3-4.7) lower absolute dense volume. The inverse association was seen for any type of physical activity among women with <3.0% TC 10-year risk, but only for total and vigorous activities among women with 3.0-4.9% TC risk, and only for vigorous activity among women with ≥5.0% TC risk. The association between total activity and absolute dense volume was modified by the TC breast cancer risk (P interaction = 0.05). As anticipated, high physical activity was also associated with lower non-dense volume. No consistent association was found between physical activity and percent dense volume. CONCLUSIONS: Our results suggest that physical activity may decrease breast cancer risk through reducing mammographic density, and that the physical activity needed to reduce mammographic density may depend on background risk of breast cancer.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4403929/

Record 1184

pmcid: PMC4688876

pmid: 26715960

doi: 10.4314/mmj.v27i3.11

title: Breast enlargement in Malawian males on the standard first-line antiretroviral therapy regimen: Case reports and review of the literature.

authors

- Kwekwesa A
- Kandionamaso C
- Winata N
- Mwinjiwa E
- Joshua M
- Garone D
- Bedell R
- van Oosterhout JJ

journal: Malawi medical journal : the journal of Medical Association of Malawi

journalAbbr: Malawi Med J

year: 2015

pubdate: 2015 Sep

volume: 27

issue: 3

pages: 115-7

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4688876/

Record 1185

pmcid: PMC4508642

pmid: 26266011

doi: 10.4081/rt.2015.5765

title: Breast Metastasis from Medullary Thyroid Carcinoma in a Male Patient: Case Report and Review of the Literature.

authors

- Mandanas S
- Margaritidou E
- Christoforidou V
- Karoglou E
- Geranou C
- Chrisoulidou A
- Boudina M
- Georgopoulos K
- Pazaitou-Panayiotou K

journal: Rare tumors

journalAbbr: Rare Tumors

year: 2015

pubdate: 2015 May 5

volume: 7

issue: 2

pages: 5765

abstract: Medullary thyroid carcinoma (MTC) is a rare malignancy that may metastasize to liver, lungs and bones. Breast is an unusual metastatic site for MTC and only 20 female cases have been reported in the literature. We present a male patient in whom histological examination and immunohistochemistry of a breast mass were indicative of breast metastasis from MTC. A 67-year-old man with recent diagnosis of MTC and metastases to cervical and upper mediastinum lymph nodes was referred to our department for further treatment. At first evaluation, diagnostic imaging techniques showed lung and bone metastases and three months later the presence of liver metastases. Due to the extension of the disease, treatment with vandetanib was decided, but serious adverse events led to its interruption after two weeks. During follow-up, patient developed a painful swelling in the right breast. Ultrasound and mammography showed the presence of multiple masses to the right breast suspicious for malignancy. Core needle biopsy and histological examination of the specimen confirmed the presence of metastatic MTC. Palliative external beam irradiation was used to relieve local pain and, after one month, the patient died. Consequently, breast masses should be cautiously evaluated, mainly in the presence of a known primary malignancy. Histological and/or cytopathological examination are requisite diagnostic tools, while external beam irradiation and tyrosine kinase inhibitors may be used as palliative therapies in the concurrent presence of breast metastases from MTC.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4508642/

Record 1186

pmcid: PMC4319262

pmid: 25729684

doi: 10.4103/2230-8210.149316

title: Challenges and controversies in diagnosis and management of gonadotropin dependent precocious puberty: An Indian perspective.

authors

- Kumar M
- Mukhopadhyay S
- Dutta D

journal: Indian journal of endocrinology and metabolism

journalAbbr: Indian J Endocrinol Metab

year: 2015

pubdate: 2015 Mar-Apr

volume: 19

issue: 2

pages: 228-35

abstract: Managing precocious puberty (PP) has been a challenge due to lack of standardized definition, gonadotrophins assay, gonadotrophin stimulation, timings for blood sampling, and parameters for assessing outcomes. This review evaluated available literature to simplify the algorithm for managing gonadotrophin dependent/central PP (CPP), with an Indian perspective. CPP is one of the commonest forms of PP and mimics the normal course of puberty, at an age <8 and 9 years for girls and boys respectively. Basal and post gonadotrophin hormone releasing hormone analog (GnRHa) luteinizing hormone (LH) ≥0.3-0.6 IU/L and ≥4-5 IU/L (30-60 min after GnRH/GnRHa administration) respectively, using modern ultrasensitive automated chemiluminescence assays, can be considered positive for central puberty initiation. Uterine length of >3.5 cm and uterine volume of >1.8 ml are two most specific indicators for true CPP. Therapy is indicated in children with CPP with accelerated bone age, height advancement, or psychosocial stress. Treatment goal is to halt puberty progression to a socially acceptable age, allowing the child to attain optimal height potential. GnRHa is the treatment of choice, with best height outcomes when initiated <6 years age. Treatment is recommended till 11 years age. LH suppression to <3 U/L may be a reasonable target in patients on GnRHa therapy. Medroxyprogesterone acetate holds an important place in managing PP in India, cause of high costs associated with GnRHa therapy. There is an urgent need for clinical trials from India, for establishing Indian cut-off for diagnosis, treatment and follow-up of children with PP.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4319262/

Record 1187

pmcid: PMC4804665

pmid: 27057392

doi: 10.4103/2141-9248.177974

title: Considerations in Management of Rhegmatogenous Retinal Detachment in One Eyed Pregnant Females: A Report of Two Cases.

authors

- Ashish M

- Alok S
- Elesh J
- Shubhi T

journal: Annals of medical and health sciences research

journalAbbr: Ann Med Health Sci Res

year: 2015

pubdate: 2015 Nov-Dec

volume: 5

issue: 6

pages: 480-2

abstract: The aim of this case report is to discuss issues related to management of one eyed pregnant female. A 26-year-old female (Patient A) and 28-year-old female (Patient B) both in second trimester and one eyed presented to our outpatient department with diminution of vision due to rhegmatogenous retinal detachment (RRD). Retina was attached in Patient A following scleral buckling surgery but Patient B required pars plana vitrectomy with silicon oil tamponade. Best corrected visual acuity in both patients did improve from 1/60 to 6/24 and 6/18, respectively at 6 months follow-up. Exudative RDs are known to occur in pregnancy as a complication of preeclampsia, but RRD in pregnancy although co-incidental poses certain challenge with regard to management of such cases especially if the patient is one-eyed. Things to consider for management include (1) type of anesthesia (2) surgical positioning (3) positioning after surgery (4) anti-glaucoma medication if required (5) corticosteroid treatment in pregnancy (6) to provide them ambulatory vision as early as possible. With proper management and monitoring it is possible to provide them with early ambulatory vision without offering any harm to her pregnancy and fetus.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4804665/

Record 1188

pmcid: PMC4352492

pmid: 25793137

doi: 10.1155/2015/601904

title: Delayed total breast reconstruction with brava.

authors

- Hammer-Hansen N
- Jensen TB
- Damsgaard TE

journal: Case reports in surgery

journalAbbr: Case Rep Surg

year: 2015

pubdate: 2015

volume: 2015

issue:

pages: 601904

abstract: Several surgical procedures exist in regard to reconstruction of the breast after mastectomy. The use of Brava a vacuum-based external soft-tissue expansion system in combination with fat transplantation is a less documented but viable option in treating patients after mastectomy. We share our experience in treating a 57-year-old patient with mastectomy, describing the complications and pitfalls we experienced when using the

Brava system in combination with fat transplantation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4352492/

Record 1189

pmcid: PMC4366769

pmid: 25932386

doi: 10.4103/2230-8210.152764

title: Feminizing adrenocortical tumors: Literature review.

authors

- Chentli F
- Bekkaye I
- Azzoug S

journal: Indian journal of endocrinology and metabolism

journalAbbr: Indian J Endocrinol Metab

year: 2015

pubdate: 2015 May-Jun

volume: 19

issue: 3

pages: 332-9

abstract: Feminizing adrenal tumors (FAT) are extremely rare tumors prevailing in males. Clinical manifestations are gynecomastia and/or other hypogonadism features in adults. They are rarer in pediatric population and their main manifestation is peripheral sexual precocity. In women genital bleeding, uterus hypertrophy, high blood pressure and/or abdomen mass may be the only manifestations. On the biological point, estrogen overproduction with or without increase in other adrenal hormones are the main abnormalities. Radiological examination usually shows the tumor, describes its limits and its eventual metastases. Adrenal and endocrine origins are confirmed by biochemical assessments and histology, but that one is unable to distinguish between benign and malignant tumors, except if metastases are already present. Immunostaining using anti-aromatase antibodies is the only tool that distinguishes FAT from other adrenocortical tumors. Abdominal surgery is the best and the first line treatment. For large tumors (≥10 cm), an open access is preferred to coeliosurgery, but for the small ones, or when the surgeon is experienced, endoscopic surgery seems to give excellent results. Surgery can be preceded by adrenolytic agents such as ortho paraprime dichloro diphenyl dichloroethane (Mitotane), ketoconazole or by aromatase inhibitors, but till now there is not any controlled study to compare the benefit of different drugs. New anti-estrogens can be used too, but their results need to be confirmed in malignant tumors resistant to classical chemotherapy and to conventional radiotherapy. Targeted therapy can be used too, as in other adrenocortical tumors, but the results need to be confirmed.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4366769/

Record 1190

pmcid: PMC4434366

pmid: 25948845

doi: 10.1136/bcr-2014-207474

title: Fluoxymesterone-induced gynaecomastia in a patient with childhood aplastic anaemia.

authors

- Lo TE
- Andal ZC

- Lantion-Ang FL

journal: BMJ case reports

journalAbbr: BMJ Case Rep

year: 2015

pubdate: 2015 May 6

volume: 2015

issue:

pages:

abstract: Gynaecomastia is a benign condition characterised by enlargement of the male breast. Drug-induced gynaecomastia merits deep consideration as it may account for as many as 25% of all cases of gynaecomastia in adults. Although the mechanism is not fully clear, some mechanisms include oestrogen-like activities, stimulation of testicular production of oestrogens, inhibition of testosterone synthesis or blockade of androgen action. Anabolic steroids, in particular when used during the pubertal stage, may cause significant irreversible gynaecomastia. We report a case of 28-year-old Filipino man with persistent gynaecomastia from fluoxymesterone used for aplastic anaemia during his prepubertal stage. Hormonal work ups for gynaecomastia all turned out normal, thus isolating the drug as the cause. The patient was unable to undergo breast reconstruction surgery due to haematological contraindications, but eventually referred to psychiatry for counselling. This case will highlight the paradoxical effect of androgenic steroid used during childhood on male breast proliferation during puberty.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4434366/

Record 1191

pmcid: PMC4374859

pmid: 25811990

doi: 10.1371/journal.pone.0122279

title: Genotype-phenotype correlation in Chinese patients with spinal and bulbar muscular atrophy.

authors

- Ni W
- Chen S
- Qiao K
- Wang N
- Wu ZY

journal: PloS one

journalAbbr: PLoS One

year: 2015

pubdate: 2015

volume: 10

issue: 3

pages: e0122279

abstract: Spinal and bulbar muscular atrophy (SBMA) is an X-linked recessive motor neuron disease characterized by slowly progressive weakness and atrophy of proximal limbs and bulbar muscles. To assess the genotype-phenotype correlation in Chinese patients, we identified 155 patients with SBMA and retrospectively examined available data from laboratory tests and neurophysiological analyses. Correlations between genotype and phenotype were analyzed. There was an inverse correlation between the length of CAG repeats and age at

first muscle weakness (p<0.0001). The serum creatine kinase level showed a significant inverse correlation with disease duration and the age at examination (p=0.019 and p=0.004, respectively). Unlike previous classification of motor- and sensory-dominant phenotypes, all findings of nerve conduction, except the amplitudes of median nerve compound motor action potential, were positively correlated to the length of CAG repeats. A significant decline in sensory nerve action potential amplitudes may assist differential diagnosis of SBMA.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4374859/

Record 1192

pmcid: PMC4680110

pmid: 26713166

doi: 10.1155/2015/892369

title: Gestational Gigantomastia Complicating Pregnancy: A Case Report and Review of the Literature.

authors

- Rezai S
- Nakagawa JT
- Tedesco J
- Chadee A
- Gottimukkala S
- Mercado R
- Henderson CE

journal: Case reports in obstetrics and gynecology

journalAbbr: Case Rep Obstet Gynecol

year: 2015

pubdate: 2015

volume: 2015

issue:

pages: 892369

abstract: Background. Gestational gigantomastia is a rare disorder without clear etiology or well-established risk factors. Several pathogenic mechanisms contributing to the disease process have been proposed, all of which can lead to a similar phenotype of breast hypertrophy. Case. A 28-year-old Guinean woman presented at 37 weeks of gestation with bilateral gigantomastia, mastalgia, peau d'orange, and back pain. Prolactin levels were 103.3 μg/L (with a normal reference value for prolactin in pregnancy being 36-372 μg/L). The patient was treated with bromocriptine (2.5 mg twice daily), scheduled for a repeat cesarean, and referred to surgery for bilateral mammoplasty. Conclusion. Gestational gigantomastia is a rare disorder, characterized by enlargement and hypertrophy of breast tissue. Our patient presented with no endocrine or hematological abnormalities, adding to a review of the literature for differential diagnoses, workup, and management of cases of gestational gigantomastia with normal hormone levels.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4680110/

Record 1193

pmcid: PMC4412434

pmid: 25827472

doi: 10.1007/s00228-015-1835-x

title: Gynecomastia and drugs: a critical evaluation of the literature.

authors

- Nuttall FQ
- Warrier RS
- Gannon MC

journal: European journal of clinical pharmacology

journalAbbr: Eur J Clin Pharmacol

year: 2015

pubdate: 2015 May

volume: 71

issue: 5

pages: 569-78

abstract: PURPOSE: A large number of medications have been implicated in the genesis of gynecomastia. However, gynecomastia is common in men, asymptomatic, increases with age, and is considered to be due to an increased estradiol/testosterone ratio. This complicates the interpretation of medication-related gynecomastia. Therefore, we have reviewed the literature in order to assess the data relating gynecomastia onset with utilization of specific medications. METHODS: The literature was searched in PubMed and the Ovid/Medline databases from the 1946 to January 2015 with the search terminology of "gynecomastia, drugs/medications." A few other articles were found and included. RESULTS: One hundred ten publications were reviewed. Sixty-three were single case reports. There were 24 population-based studies of which 8 were HIV-infected patients treated with antiretroviral agents. Among the case reports, 49 were for individual medications, and 2 were reports of antineoplastic or antiretroviral drug regimens. In the great majority, mastodynia with or without breast enlargement was present and referred to as gynecomastia. Generally, hormonal profiles could not explain the breast enlargement. The pain/tenderness and breast enlargement resolved spontaneously over time. CONCLUSION: Many different medications have been associated with the presence of "gynecomastia." Generally, it presents as a syndrome characterized by a single painful/tender breast (mastodynia) associated with breast enlargement and is transient. We suggest that these cases be referred to as an acute gynecomastia syndrome. This syndrome also occurs independent of medication use. Thus, in an individual patient, whether it is medication induced often remains uncertain. The pathogenesis remains unknown.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4412434/

Record 1194

pmcid: PMC4521169

pmid: 26240797

doi: 10.5415/apallergy.2015.5.3.187

title: Gynecomastia induced by H1-antihistamine (ebastine) in a patient with idiopathic anaphylaxis.

authors

- Jung HS
- Park CH
- Park YT
- Bae MA
- Lee YI
- Kang BJ
- Jegal Y
- Ahn JJ
- Lee T

journal: Asia Pacific allergy

journalAbbr: Asia Pac Allergy

year: 2015

pubdate: 2015 Jul

volume: 5

issue: 3

pages: 187-90

abstract: H1-antihistamine is generally a well-tolerated and safe drug. However, in resemblance with all other drugs, H1-antihistamines can also prompt adverse drug reactions (ADRs). We recently encountered the very unusual ADR of H1-antihistamine-induced gynecomastia. A 21-year-old man with idiopathic anaphylaxis was treated with ebastine (Ebastel), a second-generation H1-antihistamine, for the prevention of anaphylaxis. Three months later, the patient remained well without anaphylaxis, but had newly developed gynecomastia. Because anaphylaxis recurred after the cessation of H1-antihistamine, the preventive medication was changed to omalizumab. A few months later, his gynecomastia had entirely disappeared. Physicians should be aware of this exceptional ADR of H1-antihistamine.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4521169/

Record 1195

pmcid: PMC4687699

pmid: 26416346

doi: 10.1111/jcmm.12673

title: In vitro expression of cytokeratin 18, 19 and tube formation of adipose-derived stem cells induced by the breast epithelial cell line HBL-100.

authors

- Yang J
- Xiong L
- Wang R
- Yuan Q
- Xia Y
- Sun J
- Horch RE

journal: Journal of cellular and molecular medicine

journalAbbr: J Cell Mol Med

year: 2015

pubdate: 2015 Dec

volume: 19

issue: 12

pages: 2827-31

abstract: Fat transplantation is increasingly used in breast augmentation; and recently, the issue of safety concerns from a cellular and molecular point of view has been raised. In this study, attentions were paid to the interaction between adipose-derived stem cells (ADSC) and mammary epithelial cells: human breast cancer cell line - 100 (HBL - 100) cells were used to simulate the normal microenvironment in breast tissue, ADSCs were harvest from human and co-cultured with HBL-100 cells. It was found that ADSCs formed tube-like structures in the co-culture with HBL-100 cells in contrast to the normal morphology of ADSCs in the control group. In

addition, the immunofluorescence imaging showed that cytokeratin 18 and 19 (CK18 and 19) were significantly expressed in ADSCs after the co-culture with HBL-100 cells. The ultrastructure of those ADSCs also showed epithelial changes. In conclusion, ADSCs are not biological stable when co-cultured with HBL-100 cells. They differentiate into epithelial-like cells with the expression of epithelial surface marks (CK 18, 19) and form tube-like structures. This may offer an important evidence for the further study of clinical application of transplanting ADSCs rich adipose tissue into the breast in the future.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4687699/

Record 1196

pmcid: PMC4673580

pmid: 26394847

doi: 10.3109/13625187.2015.1074675

title: Inhibition of ovulation by administration of estetrol in combination with drospirenone or levonorgestrel: Results of a phase II dose-finding pilot study.

authors

- Duijkers IJ
- Klipping C
- Zimmerman Y
- Appels N
- Jost M
- Maillard C
- Mawet M
- Foidart JM
- Coelingh Bennink HJ

journal: The European journal of contraception & reproductive health care : the official journal of the European Society of Contraception

journalAbbr: Eur J Contracept Reprod Health Care

year: 2015

pubdate: 2015

volume: 20

issue: 6

pages: 476-89

abstract: OBJECTIVES: The aim of the study was to evaluate the efficacy of different dosages of estetrol (E4) combined with one of two progestins in suppressing the pituitary-ovarian axis and ovulation in healthy premenopausal women. METHODS: This was an open, parallel, phase II, dose-finding, pilot study performed in healthy women aged 18 to 35 years with a documented ovulatory cycle before treatment. For three consecutive cycles in a 24/4-day regimen, participants received 5 mg or 10 mg E4/3 mg drospirenone (DRSP); 5 mg, 10 mg or 20 mg E4/150 µg levonorgestrel; or 20 µg ethinylestradiol (EE)/3 mg DRSP as comparator. Pituitary-ovarian axis activity and the occurrence of ovulation were evaluated by monitoring follicular size, serum levels of follicle-stimulating hormone, luteinising hormone, estradiol and progesterone during treatment cycles 1 and 3. Endometrial thickness was evaluated throughout the trial, and the return of ovulation was evaluated after the last intake of medication. RESULTS: A total of 109 women were included in the trial. No ovulation occurred in any treatment group. Ovarian activity inhibition seemed proportional to the E4 dosage: the highest suppression was observed in the 20 mg E4 group and was very similar to that observed with EE/DRSP. Endometrial thickness was suppressed to the same extent in all groups. Post-treatment ovulation occurred in all participants between 17 and 21 days after the last active treatment. The study combinations were well tolerated and safe. CONCLUSIONS: Combined with a progestin, E4 adequately suppresses ovarian activity, particularly when

given at a dosage above 10 mg/day.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4673580/

Record 1197

pmcid: PMC4764134

pmid: 26918066

doi: 10.1080/19317611.2014.999967

title: Internet Pornography Use and Sexual Body Image in a Dutch Sample.

authors

- Cranney S

journal: International journal of sexual health : official journal of the World Association for Sexual Health

journalAbbr: Int J Sex Health

year: 2015

pubdate: 2015

volume: 27

issue: 3

pages: 316-323

abstract: OBJECTIVES: A commonly attributed cause of sexual body image dissatisfaction is pornography use. This relationship has received little verification. METHODS: The relationship between sexual body image dissatisfaction and Internet pornography use was tested using a large-N sample of Dutch respondents. RESULTS/CONCLUSION: Penis size dissatisfaction is associated with pornography use. The relationship between pornography use and breast size dissatisfaction is null. These results support prior speculation and self-reports about the relationship between pornography use and sexual body image among men. These results also support a prior null finding of the relationship between breast size satisfaction for women and pornography use.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4764134/

Record 1198

pmcid: PMC5351447

pmid: 28331703

doi: 10.5152/tjbh.2015.2565

title: Mammography Findings of Male Breast Diseases.

authors

- Şafak KY

journal: The journal of breast health

journalAbbr: J Breast Health

year: 2015

pubdate: 2015 Jul

volume: 11

issue: 3

pages: 106-110

abstract: Over the past 2 decades, the percentage of men presenting with breast complaints has increased from

0.8% to 2.4%, and men now account for 1% of all breast cancer cases. The most common male breast mass is gynecomastia, followed by lipoma and epidermal inclusion cysts. Because there is a paucity of parenchyma as compared with the female breast, the malignancy rapidly progresses to the next stage, with the appearance of secondary signs like nipple retraction, fixation to deeper tissues, skin ulceration or adenopathy. Diagnostic evaluation is needed only when the palpable mass is unilateral, hard, fixed, peripheral to the nipple, or associated with nipple discharge, skin changes, or lymphadenopathy. Male breast cancer usually occurs in a subareolar location or is positioned eccentric to the nipple; occasionally, it occurs in a peripheral position. Secondary signs like skin thickening, nipple retraction, and axillary lymphadenopathy may be seen. Microcalcifications can occur. Mammography can accurately distinguish between malignant and benign male breast disease. Radiologists are generally less familiar with breast disease in males compared with females. In this article, we discuss the clinical, and mammographic features of a variety of benign and malignant diseases that can occur in the male breast.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5351447/

Record 1199

pmcid: PMC4452962

pmid: 26011195

doi: 10.9738/INTSURG-D-14-00231.1

title: Management of granulomatous mastitis: a series of 13 patients who were evaluated for treatment without corticosteroids.

authors

- Yukawa M
- Watatani M
- Isono S
- Fujiwara Y
- Tsujie M
- Kitani K
- Hara J
- Kato H
- Takeyama H
- Kanaizumi H
- Kogata S
- Ohta Y
- Inoue M

journal: International surgery

journalAbbr: Int Surg

year: 2015

pubdate: 2015 May

volume: 100

issue: 5

pages: 774-82

abstract: Granulomatous mastitis (GM) is a rare chronic inflammatory breast condition with unknown etiology. There is still no generally accepted optimal treatment for GM. Corticosteroid treatment and/or wide excision is most commonly reported in the literature. Incision and drainage or limited excision alone has little benefit because of a strong tendency of recurrence. Corticosteroids also have a high failure rate and possible side

effects. In the current series, we treated GM patients without corticosteroids, except for one patient. We also devised multidirectional deep drainage for advanced and complicated abscesses, which are characteristic of GM. This retrospective study included 13 women who met the required histologic criteria of GM. The mean age of the patients was 41 years. All of the patients were premenopausal. Six patients had breast-fed in the last 5 years. Five patients were under medication with antidepressants. A total of 11 patients developed abscesses during the clinical course, and the abscesses penetrated the retromammary space in 4 patients. We treated 2 of these 4 patients with multidirectional deep drainage and obtained complete remission in 5 and 6.5 months, respectively. These times were much shorter than those in the other 2 patients. The time to resolution in 11 patients was 4 to 28 months. This overall outcome was comparable with that of corticosteroid treatment reported in the literature. Because the natural history of GM is thought to be self-limiting, close observation and minimally required drainage of abscesses without corticosteroid administration remain the treatment modality of choice.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4452962/

Record 1200

pmcid: PMC4701750

pmid: 26610198

doi: 10.1016/j.ijscr.2015.10.008

title: Management of pregnancy associated breast cancer with chemotherapy in a developing country.

authors

- Sule EA
- Ewemade F

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2015

pubdate: 2015

volume: 17

issue:

pages: 117-20

abstract: CONTEXT: Although breast cancer is a common cancer, Pregnancy associated breast cancer is uncommon. Adjuvant chemotherapy administered intrapartum has been resolved to be safe from the second trimester. OBJECTIVE: To review cases of pregnancy associated breast cancer managed with adjuvant intrapartum chemotherapy. PATIENTS AND METHOD: Gravid patients diagnosed with breast cancer had chemotherapy administered by a slow infusion protocol at 3 weekly interval from the second trimester till 4 weeks to expected date of delivery. Obstetric scans were done to monitor fetal growth and development. Requisite surgery was carried out intrapartum and postpartum. RESULTS: There were three cases of pregnancy associated breast cancer Age range 32-33 years, mean 32.5 years. Two cases presented in the second trimester while one presented in the third trimester. The second case had overt metastatic disease and was grave in respiratory distress. Histology showed invasive lobular carcinoma in two cases and extensive intraductal carcinoma with invasive component in the third. Immunohistochemistry showed triple negative in the first case and hormone positive in the third case. Wide local excision was done for a 3cm lump in the first case and mastectomy postpartum in the third case. but had no surgery. Doxorubicin and cyclophosphamide was administered three weekly from the second trimester up to 32 weeks and continued postpartum. Taxanes was administered afterwards. The grave clinical state of the second case was markedly improved with the first cycle of chemotherapy instituted. All cases had spontaneous vaginal delivery with good apgar scores. Children had normal developmental milestones. First case with breast conservation is clinically disease free, the second case demised postpartum from disease progression while the third had a mastectomy and is on cue for radiotherapy. CONCLUSION: Adjuvant intrapartum chemotherapy had a successful outcome with birth of normal babies with normal developmental milestones in our miniseries.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4701750/

Record 1201

pmcid: PMC4727687

pmid: 26894003

doi: 10.1097/GOX.0000000000000564

title: Mechanobiology and Mechanotherapy of Adipose Tissue-Effect of Mechanical Force on Fat Tissue Engineering.

authors

- Yuan Y
- Gao J
- Ogawa R

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2015

pubdate: 2015 Dec

volume: 3

issue: 12

pages: e578

abstract: Our bodies are subjected to various mechanical forces, which in turn affect both the structure and function of our bodies. In particular, these mechanical forces play an important role in tissue growth and regeneration. Adipocytes and adipose-derived stem cells are both mechanosensitive and mechanoresponsive. The aim of this review is to summarize the relationship between mechanobiology and adipogenesis. PubMed was used to search for articles using the following keywords: mechanobiology, adipogenesis, adipose-derived stem cells, and cytoskeleton. In vitro and in vivo experiments have shown that adipogenesis is strongly promoted/inhibited by various internal and external mechanical forces, and that these effects are mediated by changes in the cytoskeleton of adipose-derived stem cells and/or various signaling pathways. Thus, adipose tissue engineering could be enhanced by the careful application of mechanical forces. It was shown recently that mature adipose tissue regenerates in an adipose tissue-engineering chamber. This observation has great potential for the reconstruction of soft tissue deficiencies, but the mechanisms behind it remain to be elucidated. On the basis of our understanding of mechanobiology, we hypothesize that the chamber removes mechanical force on the fat that normally impose high cytoskeletal tension. The reduction in tension in adipose stem cells triggers their differentiation into adipocytes. The improvement in our understanding of the relationship between mechanobiology and adipogenesis means that in the near future, we may be able to increase or decrease body fat, as needed in the clinic, by controlling the tension that is loaded onto fat.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4727687/

Record 1202

pmcid: PMC4789950

pmid: 26989360

doi: 10.1159/000441288

title: Metaplastic Breast Carcinoma with Unusual Presentation: Review of Three Cases.

authors

- Fernández Pérez MA
- Viqueira Rodriguez I

- Tello Royloa A
- Martínez Guisasola J

journal: Breast care (Basel, Switzerland)

journalAbbr: Breast Care (Basel)

year: 2015

pubdate: 2015 Dec

volume: 10

issue: 6

pages: 404-7

abstract: BACKGROUND: Metaplastic breast carcinoma is an uncommon type of breast cancer that usually appears as a large, fast growing breast lump. CASE REPORT: We report 3 cases of metaplastic breast carcinoma presented at our clinic in 2014. The mean age at diagnosis was 67.3 years. 1 patient presented with a fast growing, large mass detected by herself. However, in the other 2 patients, the nodule was approximately 1.5 cm in size, not fast growing, and was detected on mammography. All 3 patients were treated surgically (2 lumpectomies and 1 mastectomy), with the final pathology of metaplastic carcinoma with chondroid mesenchymal differentiation in 2 cases and metaplastic carcinoma with myoepithelial differentiation in the 3rd case. The patients are still under adjuvant therapy. CONCLUSIONS: Metaplastic breast carcinoma may present unusually as a non-palpable lump. This entity must be considered in any breast lump.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4789950/

Record 1203

pmcid: PMC4613638

pmid: 26318471

doi: 10.15537/smj.2015.9.11778

title: Mixed gynecomastia.

authors

- Al Qassabi SS
- Al-Harthi SM
- Al-Osali ME

journal: Saudi medical journal

journalAbbr: Saudi Med J

year: 2015

pubdate: 2015 Sep

volume: 36

issue: 9

pages: 1115-7

abstract: Gynecomastia is an enlargement of male breast resulting from a proliferation of its glandular component, and it is usually due to an altered estrogen-androgen balance. It should be differentiated from pseudogynecomastia, which is characterized by fat deposition without glandular proliferation and from breast carcinoma. Gynecomastia could be physiological in neonates and pubertal or pathological due to drug intake, chronic liver, or renal disease, hyperthyroidism, testicular or adrenal neoplasms, and hypogonadism whether primary, or secondary. Properly organized work-up is needed to reach the cause of gynecomastia. Here, we reported a case of a young Omani man with gynecomastia with the aim of creating awareness of the occurrence of Klinefelter's syndrome (KS) in patients with gynecomastia, to observe any differences in clinical presentation of KS from those reported in the literature, and highlight the needed diagnostic work-up and treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4613638/

Record 1204

pmcid: PMC4361163

pmid: 25797961

doi: 10.1055/s-0034-1383418

title: Multifocal Tumorous Pseudoangiomatous Stromal Hyperplasia Presenting as Asymmetric Bilateral Breast Enlargement.

authors

- Vasconcelos I
- Perez Fernandez CM
- Günzel S
- Schoenegg W

journal: Geburtshilfe und Frauenheilkunde

journalAbbr: Geburtshilfe Frauenheilkd

year: 2015

pubdate: 2015 Feb

volume: 75

issue: 2

pages: 183-187

abstract: PASH is a benign proliferation of stromal myofibroblasts that affects mostly premenopausal women and typically shows estrogen and progesterone receptor expression, allowing speculation regarding a hormonal cause. It usually presents as an incidental finding on a mammogram or as a palpable mass. We present a case of diffuse asymmetrical massive breast enlargement in a premenopausal woman with history of previous multiple PASH excisions for recurrent lesions, caused by multifocal tumorous PASH virtually replacing the breast parenchyma. Immunohistochemistry examination showed no hormone receptor expression. Despite its benign nature, such presentation of PASH is managed with bilateral mastectomy and immediate reconstruction with expanders for cosmetic and comfort reasons, while tumor excision or expectant management is deemed to lead to recurrence and progression. Although a hormonal origin is speculated based on hormone expression studies and typical patient profile, this case showed 0 % estrogen/progesterone expression in the final histology specimen.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4361163/

Record 1205

pmcid: PMC4439227

pmid: 26029298

doi: 10.5489/cuaj.2367

title: Multiple urethral anomalies: Parameatal urethral cyst, penile curvature, incomplete hypospadiac anterior duplication of the urethra and distal hipospadias.

authors

- Tuglu D
- Yuvanç E
- Yılmaz E
- Gur S

- Batislam E

journal: Canadian Urological Association journal = Journal de l'Association des urologues du Canada

journalAbbr: Can Urol Assoc J

year: 2015

pubdate: 2015 May-Jun

volume: 9

issue: 5-6

pages: E294-6

abstract: The male genitourinary system is quite complex. There are numerous known anomalies of the male urethra either as isolated cases or in combination with other disorders. An improved understanding of the embryology and anatomy of the normal male urethral development might help explain the causes of the various urethral abnormalities. We contribute to the etiology of congenital anomalies with this multiple urethral anomalies case.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4439227/

Record 1206

pmcid: PMC4678666

pmid: 26666878

doi: 10.1186/s12902-015-0076-4

title: Optic glioma and precocious puberty in a girl with neurofibromatosis type 1 carrying an R681X mutation of NF1: case report and review of the literature.

authors

- Kocova M
- Kochova E
- Sukarova-Angelovska E

journal: BMC endocrine disorders

journalAbbr: BMC Endocr Disord

year: 2015

pubdate: 2015 Dec 15

volume: 15

issue:

pages: 82

abstract: BACKGROUND: Neurofibromatosis type 1 (NF1) is a common autosomal dominant genetic disorder with an extremely variable phenotype. In childhood NF1 can be associated with optic glioma and central precocious puberty; the latter is more common when the optic chiasm is affected. The mutational spectrum of the NF1 gene is wide and complex; R681X is a rare severe mutation of the NF1 gene known to cause truncation of neurofibromin, with only ten reported cases in the literature so far. CASE PRESENTATION: We describe a girl with NF1 associated with early central precocious puberty appearing at 2.5 years of age and optic glioma affecting the optic chiasm as seen on magnetic resonance imaging (MRI). Genetic analysis confirmed the presence of R681X. Therapy with a gonadotropin-releasing hormone agonist was instituted with good response to therapy. The lesions on MRI were stable and no significant vision impairment was present during the 6 years of follow-up. CONCLUSION: Of the ten reported cases of NF1 due to R681X, one has presented with optic glioma and none with precocious puberty. Thus, to our knowledge, this is the first reported case of this mutation presenting with precocious puberty. We believe that this is a contribution to the few reports on the phenotype of this mutation and to the future elucidation of genotype-phenotype correlations of this disease.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4678666/

Record 1207

pmcid: PMC4672610

pmid: 26793446

doi: 10.1016/j.idcr.2014.12.004

title: Osteomyelitis of sternum and rib after breast prosthesis implantation: A rare or underestimated infection?

authors

- Seng P
- Alliez A
- Honnorat E
- Menard A
- Casanova D
- Stein A

journal: IDCases

journalAbbr: IDCases

year: 2015

pubdate: 2015

volume: 2

issue: 1

pages: 31-3

abstract: Sternum and rib osteomyelitis complicated from breast implant infection is rare. We report a case of early sternum and rib osteomyelitis occurred during breast implant infection managed in an inter-regional referral center for bone/joint infections in the south of France.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4672610/

Record 1208

pmcid: PMC4606588

pmid: 26539378

doi: 10.4103/2229-516X.165370

title: Para-meatal urethral cyst with bladder calculi: A case report and review of the literature.

authors

- Patil SB
- Patil NA
- Kundargi VS
- Biradar AN

journal: International journal of applied & basic medical research

journalAbbr: Int J Appl Basic Med Res

year: 2015

pubdate: 2015 Sep-Dec

volume: 5

issue: 3

pages: 220-1

abstract: Para-meatal urethral cyst or median raphe cyst of the glans penis is a benign cystic lesion uncommonly encountered in prepubertal age group. Approximately, about 50 cases have been reported till date in the world literature. Herein we report a case of para-meatal urethral cyst concurrent with urinary bladder calculi.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4606588/

Record 1209

pmcid: PMC4508269

pmid: 25433660

doi: 10.3803/EnM.2015.30.2.226

title: Partial Androgen Insensitivity Syndrome Presenting with Gynecomastia.

authors

- Lee SW
- Kwak DS
- Jung IS
- Kwak JH
- Park JH
- Hong SM
- Lee CB
- Park YS
- Kim DS
- Choi WH
- Ahn YH

journal: Endocrinology and metabolism (Seoul, Korea)

journalAbbr: Endocrinol Metab (Seoul)

year: 2015

pubdate: 2015 Jun

volume: 30

issue: 2

pages: 226-30

abstract: Gynecomastia is a benign enlargement of the male breast caused by the proliferation of glandular breast tissue. Determining the various causes of gynecomastia such as physiological causes, drugs, systemic diseases, and endocrine disorders is important. Androgen insensitivity syndrome (AIS) is a rare endocrine disorder presenting with gynecomastia and is a disorder of male sexual differentiation caused by mutations within the androgen receptor gene. All individuals with AIS have the 46 XY karyotype, although AIS phenotypes can be classified as mild, partial or complete and can differ among both males and females including ambiguous genitalia or infertility in males. We experienced a case of partial AIS presenting with gynecomastia and identified the androgen receptor gene mutation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4508269/

Record 1210

pmcid: PMC4478398

pmid: 26137191

doi: 10.4314/mmj.v27i1.3

title: Pathologically confirmed breast cancer in Malawi: a descriptive study: Clinical profile of breast cancer.

authors

- Kohler RE
- Moses A
- Krysiak R
- Liomba NG
- Gopal S

journal: Malawi medical journal : the journal of Medical Association of Malawi

journalAbbr: Malawi Med J

year: 2015

pubdate: 2015 Mar

volume: 27

issue: 1

pages: 10-2

abstract: BACKGROUND: Breast cancer is the most common female cancer in Africa, yet no published studies have investigated breast cancer in Malawi. Understanding the clinical profile of breast cancer is important to develop early diagnosis efforts. AIM: To describe clinical and pathological characteristics of breast specimens from a pathology laboratory at a national teaching hospital. METHODS: Secondary analysis of pathology reports from July 2011 to September 2013. RESULTS: Among 85 breast cancer cases, 55% were < 50 years. Median tumor size was 4 cm and 49% were grade 3. Median symptom duration was eight months. CONCLUSIONS: Malawian women with breast cancer commonly have long symptom durations prior to diagnosis, young age, and poorly differentiated tumors. Improved clinical and pathological characterization, including hormone receptor status, are urgently needed to better understand this disease in Malawi.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4478398/

Record 1211

pmcid: PMC4677553

pmid: 26831552

doi: 10.4274/jcrpe.1994

title: Plasma Kisspeptin Levels in Newborn Infants with Breast Enlargement.

authors

- Kaya A
- Orbak Z
- Polat H
- Çayır A
- Erdil A
- Döneray H

journal: Journal of clinical research in pediatric endocrinology

journalAbbr: J Clin Res Pediatr Endocrinol

year: 2015

pubdate: 2015 Sep

volume: 7

issue: 3

pages: 192-6

abstract: OBJECTIVE: Kisspeptin levels have been reported in children with premature thelarche, precocious puberty and adolescent gynecomastia, but there are no reports on kisspeptin levels in the neonatal period. This study aimed to investigate plasma kisspeptin hormone levels in newborns with and without breast enlargement. METHODS: Plasma kisspeptin levels and other related biochemical variables were investigated in this prospective study conducted on 40 (20 girls and 20 boys) newborn infants with breast enlargement and on 40 healthy control infants (20 girls and 20 boys). Two-milliliter venous blood samples were taken in hemogram tubes with K2EDTA. Kisspeptin assays were performed using the enzyme-immunoassay method. RESULTS: Mean plasma kisspeptin levels were 0.6 ± 0.2 ng/mL in the study group and 0.5 ± 0.2 ng/mL in the control group. Plasma kisspeptin concentrations were significantly higher in the study group (p=0.039) and also showed a correlation with serum prolactin levels (p=0.006). Significant correlations were also determined between plasma kisspeptin and luteinizing hormone concentrations (p=0.05, r=0.312). CONCLUSION: The findings of this study suggest that plasma kisspeptin and serum prolactin levels may be involved in the physiopathology of breast enlargement in newborns.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4677553/

Record 1212

pmcid: PMC4582367

pmid:

doi:

title: Polycystic Liver Disease Presenting as Expanding Mass Following Cholecystectomy.

authors

- Madigan CG
- Wang JY

journal: Hawai'i Journal of Medicine & Public Health

journalAbbr: Hawaii J Med Public Health

year: 2015

pubdate: 2015 Sep

volume: 74

issue: 9 Suppl 2

pages: 53

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4582367/

Record 1213

pmcid: PMC4582641

pmid: 26413264

doi: 10.1186/s13569-015-0028-9

title: Primary and secondary angiosarcomas: a comparative single-center analysis.

authors

- Hillenbrand T
- Menge F

- Hohenberger P
- Kasper B

journal: Clinical sarcoma research

journalAbbr: Clin Sarcoma Res

year: 2015

pubdate: 2015

volume: 5

issue:

pages: 14

abstract: BACKGROUND: Angiosarcomas (AS) are rare vascular malignancies. They are subdivided into primary (PAS) and secondary angiosarcomas (SAS). The objective was to compare the characteristics of AS subtypes. METHODS: Eighteen PAS and ten SAS patients treated at our institution between 2004 and 2012 were included in this study. RESULTS: Median age of PAS and SAS patients was 52.9 and 64.2 years, respectively (p = 0.1448). The percentage of women was 27.8% for PAS, but 80.0% for SAS (p = 0.0163). While PAS occurred throughout the body, the majority of SAS arose from the breast (p = 0.0012). All SAS were radiation-induced with a median latency of 7.7 years. The majority of patients with PAS and SAS underwent surgery as primary or recurrence treatment (p > 0.95). Local recurrence was developed by 27.8% of PAS and 50.0% of SAS (p = 0.4119). 61.1% of PAS metastasized, but only 40.0% of SAS (p = 0.4328). Median overall survival for PAS and SAS was 19 and 57 months, respectively (p = 0.2306). CONCLUSION: Radical surgery remains the mainstay of both primary and recurrence treatment. SAS show a high local recurrence rate, while PAS tend towards developing early metastases. Overall, prognosis is poor for both groups.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4582641/

Record 1214

pmcid: PMC4705094

pmid: 26770249

doi: 10.4048/jbc.2015.18.4.404

title: Primary Neuroendocrine Carcinoma of the Breast with Clinical Features of Inflammatory Breast Carcinoma: A Case Report and Literature Review.

authors

- Lee DH
- Park AY
- Seo BK
- Kim YS
- Lee KY
- Cha SH

journal: Journal of breast cancer

journalAbbr: J Breast Cancer

year: 2015

pubdate: 2015 Dec

volume: 18

issue: 4

pages: 404-8

abstract: Primary neuroendocrine carcinoma of the breast (NECB) is a very rare type of invasive breast

carcinoma. Most NECBs appear on breast imaging as solid masses of varied shapes and margins, and have worse clinical outcomes than does invasive ductal carcinoma, not otherwise specified. However, there have been no reports to date regarding NECB with features of inflammatory breast carcinoma. Here, we describe the clinical, radiol-ogic, and pathologic findings of the first reported case of primary NECB presenting as inflammatory breast carcinoma. The patient complained of diffuse right breast enlargement and erythema. Mammography identified severe breast edema and axillary lymphadenopathy. Ultrasound detected an irregular, angular, hypoechoic mass with dermal lymphatic dilatation. On magnetic resonance imaging, the mass had rim enhancement and the entire right breast showed heterogeneous enhancement with malignant kinetic features. Pathology identified the mass as a primary NECB with positive for synaptophysin, CD56, estrogen and progesterone receptors.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4705094/

Record 1215

pmcid: PMC4727695

pmid: 26894011

doi: 10.1097/GOX.0000000000000567

title: Primary Squamous Cell Carcinoma Arising from a Breast Implant Capsule.

authors

- Zomerlei TA
- Samarghandi A
- Terando AM

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2015

pubdate: 2015 Dec

volume: 3

issue: 12

pages: e586

abstract: Primary squamous cell carcinoma (SCC) of the breast comprises less than 0.1% of all breast cancers. Literature review reveals only 1 reported case of an SCC arising from the capsule of a breast implant. The authors describe, herein, a primary SCC arising from the capsule of a long-standing silicone breast implant.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4727695/

Record 1216

pmcid: PMC4677723

pmid: 26668576

doi: 10.1159/000442045

title: Prostate Cancer: Cases of Rare Presentation and Rare Metastasis.

authors

- Mandaliya H
- Sung J
- Hill J
- Samali R
- George M

journal: Case reports in oncology

journalAbbr: Case Rep Oncol

year: 2015

pubdate: 2015 Sep-Dec

volume: 8

issue: 3

pages: 526-9

abstract: Prostate cancer is the second most common cancer in men. Diagnosis of early disease is based on prostate biopsy which is carried out because of symptoms of prostatism or asymptomatic rise in PSA. On the other side, advanced disease can locally invade and metastasise to lymph nodes, bones, lungs, etc. Initial presentation of prostate cancer in form of brain metastasis is extremely seldom. Similarly, prostate cancer, which metastasised to the breast, is very rare too. Here, we discuss two unique cases of prostate cancer, one of them had an initial presentation of brain metastasis from prostate adenocarcinoma and the other case had an established diagnosis of prostate cancer metastasised to the breast. In theory, cancer can cause metastatic spread to any part of the body; however diversity into such presentation or progression from prostate cancer has not been frequently noticed.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4677723/

Record 1217

pmcid: PMC4612527

pmid: 26475873

doi: 10.1136/bcr-2014-204343

title: Pseudoangiomatous stromal hyperplasia causing massive breast enlargement.

authors

- Bourke AG
- Tiang S
- Harvey N
- McClure R

journal: BMJ case reports

journalAbbr: BMJ Case Rep

year: 2015

pubdate: 2015 Oct 16

volume: 2015

issue:

pages:

abstract: Pseudoangiomatous stromal hyperplasia (PASH) of the breast is a benign mesenchymal proliferative process, initially described by Vuitch et al. We report an unusual case of a 46-year-old woman who presented with a 6-week history of bilateral massive, asymmetrical, painful enlargement of her breasts, without a history of trauma. On clinical examination, both breasts were markedly enlarged and oedematous, but there were no discrete palpable masses. Preoperative image-guided core biopsies and surgery showed PASH. PASH is increasingly recognised as an incidental finding on image-guided core biopsy performed for screen detected lesions. There are a few reported cases of PASH presenting as rapid breast enlargement. In our case, the patient presented with painful, asymmetrical, massive breast enlargement. Awareness needs to be raised of this entity as a differential diagnosis in massive, painful breast enlargement.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4612527/

Record 1218

pmcid: PMC4596426

pmid: 26495214

doi: 10.1097/GOX.0000000000000468

title: Pseudoangiomatous Stromal Hyperplasia: A Rare Cause of Idiopathic Gigantomastia.

authors

- Roy M
- Lee J
- Aldekhayel S
- Dionisopoulos T

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2015

pubdate: 2015 Sep

volume: 3

issue: 9

pages: e501

abstract: Gigantomastia remains a rare clinical diagnosis with significant physical and psychological impacts on patients. We present the case of a 40-year-old woman with idiopathic breast enlargement. Further histological analysis of the breast tissue revealed pseudoangiomatous stromal hyperplasia. This is the first reported case of diffuse breast enlargement resulting from pseudoangiomatous stromal hyperplasia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4596426/

Record 1219

pmcid: PMC4566472

pmid: 26770146

doi:

title: Rauwolfia in the Treatment of Hypertension.

authors

- Lobay D

journal: Integrative medicine (Encinitas, Calif.)

journalAbbr: Integr Med (Encinitas)

year: 2015

pubdate: 2015 Jun

volume: 14

issue: 3

pages: 40-6

abstract: Rauwolfia serpentina is a safe and effective treatment for hypertension. The plant was used by many physicians throughout India in the 1940s and then was used throughout the world in the 1950s, including in the United States and Canada. It fell out of popularity when adverse side effects, including depression and cancer, became associated with it. This author reviews the scientific literature with regard to the use of Rauwolfia and the treatment of hypertension. The author reviews the plant's botany, chemistry, and pharmacology and

provides a researched and documented method of action for the active ingredients. With special emphasis on the plant's role in treating high blood pressure, the author looks at medical uses of the plant, critically examining its adverse side effects, toxicology, and carcinogenicity. The author refutes the association between the plant and carcinogenicity and discusses the importance of correct dosing and of screening patients to minimize the occurrence of depression. He concludes with the recommendation of use of low dose Rauwolfia (LDR) for suitable patients with hypertension. The plant provides clinicians with a safe and effective adjunct to pharmaceuticals in the treatment of high blood pressure.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4566472/

Record 1220

pmcid: PMC4648196

pmid: 25657080

doi: 10.3109/13685538.2015.1004049

title: Recommendations on the diagnosis, treatment and monitoring of hypogonadism in men.

authors

- Lunenfeld B
- Mskhalaya G
- Zitzmann M
- Arver S
- Kalinchenko S
- Tishova Y
- Morgentaler A

journal: The aging male : the official journal of the International Society for the Study of the Aging Male

journalAbbr: Aging Male

year: 2015

pubdate: 2015 Mar

volume: 18

issue: 1

pages: 5-15

abstract: Hypogonadism or Testosterone Deficiency (TD) in adult men as defined by low levels of serum testosterone accompanied by characteristic symptoms and/or signs as detailed further on can be found in long-recognized clinical entities such as Klinefelter syndrome, Kallmann syndrome, pituitary or testicular disorders, as well as in men with idiopathic, metabolic or iatrogenic conditions that result in testosterone deficiency. These recommendations do not encompass the full range of pathologies leading to hypogonadism (testosterone deficiency), but instead focus on the clinical spectrum of hypogonadism related to metabolic and idiopathic disorders that contribute to the majority of cases that occur in adult men.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4648196/

Record 1221

pmcid: PMC4382637

pmid: 25838761

doi: 10.4103/1117-6806.152719

title: Reduction Mammoplasty in a Developing Country: A 10-year Review (2001-2010) at the National Orthopaedic Hospital, Enugu.

authors

- Isiguzo C
- Ogbonnaya SI
- Udezue AO

journal: Nigerian journal of surgery : official publication of the Nigerian Surgical Research Society

journalAbbr: Niger J Surg

year: 2015

pubdate: 2015 Jan-Jun

volume: 21

issue: 1

pages: 21-5

abstract: CONTEXT: Large breast is a major problem because of associated symptomatology and aesthetic concerns. Reduction mammoplasty (RM) resolves the symptom and at the same time improves the aesthetic appearance of the breast, hence improving self-esteem and social integration. AIMS: To describe the pattern of RM in a hospital in the developing world and its impact on postgraduate surgical training. SETTINGS AND DESIGN: A retrospective review of all the RMs done in the National Orthopaedic Hospital, Enugu (a major plastic surgery training center in Nigeria) over a ten-year period (2001-2010), in the developing country of Nigeria. SUBJECTS AND METHODS: All RMs done in the hospital were reviewed after retrieving their records from operation register and medical records department. Fifteen (15) cases were retrieved and analyzed. DATA ANALYSIS: Data was analyzed with Microsoft excel 2007. RESULTS: Average age of female patients who had RM was 26.5 years and 83.3% were single. The most common complaint was abnormally large breast (macromastia). Inferior pedicle technique was commonly used. CONCLUSIONS: The results of RM are remarkable as it impact positively on the quality of life of the patients. However, the level of awareness about the availability of this service is still low in the region as shown by few cases done over the period of review and this impacts negatively on the training. The need for public awareness cannot be overemphasized.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4382637/

Record 1222

pmcid: PMC4566843

pmid: 26306727

doi: 10.1530/EC-15-0071

title: Serum adiponectin in HIV-1 and hepatitis C virus mono- and co-infected Kenyan injection drug users.

authors

- Ndombi EM
- Budambula V
- Webale MK
- Musumba FO
- Wesongah JO
- Mibei E
- Ahmed AA
- Lihana R
- Were T

journal: Endocrine connections

journalAbbr: Endocr Connect

year: 2015

pubdate: 2015 Dec

volume: 4

issue: 4

pages: 223-32

abstract: Adiponectin is an important marker of anthropometric profiles of adipose tissue. However, association of adiponectin and adiposity in HIV mono- and co-infected and hepatitis (HCV) injection drug users (IDUs) has not been elucidated. Therefore, the relationship of total adiponectin levels with anthropometric indices of adiposity was examined in HIV mono-infected (anti-retroviral treatment, ART-naive, n=16 and -experienced, n=34); HCV mono-infected, n=36; HIV and HCV co-infected (ART-naive, n=5 and -experienced, n=13); uninfected, n=19 IDUs; and healthy controls, n=16 from coastal Kenya. Anthropometric indices of adiposity were recorded and total circulating adiponectin levels were measured in serum samples using enzyme-linked immunosorbent assay. Adiponectin levels differed significantly amongst the study groups (P<0.0001). Post-hoc analyses revealed decreased levels in HIV mono-infected ART-naive IDUs in comparison to uninfected IDUs (P<0.05) and healthy controls (P<0.05). However, adiponectin levels were elevated in HCV mono-infected IDUs relative to HIV mono-infected ART-naive (P<0.001) and -experienced (P<0.001) as well as HIV and HCV co-infected ART-naive (P<0.05) IDUs. Furthermore, adiponectin correlated with weight (ρ=0.687; P=0.003) and BMI (ρ=0.598; P=0.014) in HIV mono-infected ART-naive IDUs; waist circumference (ρ=-0.626; P<0.0001), hip (ρ=-0.561; P=0.001) circumference, and bust-to-waist ratio (ρ=0.561; P=0.001) in HIV mono-infected ART-experienced IDUs; waist girth (ρ=0.375; P=0.024) in HCV mono-infected IDUs; and waist-to-hip ratio (ρ=-0.872; P=0.048) in HIV and HCV co-infected ART-naive IDUs. Altogether, these results suggest suppression of adiponectin production in treatment-naive HIV mono-infected IDUs and that circulating adiponectin is a useful surrogate marker of altered adiposity in treatment-naive and -experienced HIV and HCV mono- and co-infected IDUs.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4566843/

Record 1223

pmcid: PMC4669992

pmid: 26026134

doi: 10.1093/asj/sju156

title: Shaping of the Unaffected Breast with Brava-Assisted Autologous Fat Grafting to Obtain Symmetry after Breast Reconstruction.

authors

- Uda H
- Tomioka YK
- Sugawara Y
- Sarukawa S
- Sunaga A

journal: Aesthetic surgery journal

journalAbbr: Aesthet Surg J

year: 2015

pubdate: 2015 Jul

volume: 35

issue: 5

pages: 565-73

abstract: BACKGROUND: In breast reconstruction, symmetry is a vital issue. However, when the original breast

is unusually shaped or the patient desires augmentation at the time of reconstruction, obtaining symmetrical breasts becomes difficult. OBJECTIVES: The authors performed shaping of unaffected breasts by Brava-assisted autologous fat grafting to enhance breast symmetry, and evaluated the clinical results to validate this new approach. METHODS: Brava-assisted autologous fat grafting was performed to the unaffected breasts of 12 patients who had undergone unilateral breast reconstruction. The procedure was used for augmentation in six patients and to correct ptosis, volume, and tuberous breast deformity in three, two, and one patient, respectively. Clinical outcomes were assessed in all 12 patients. RESULTS: All patients could complete fat grafting within two sessions (one session in nine patients and two sessions in three patients). The mean volume of grafted fat per session was 211 cc in all patients. The mean retention rate of grafted fat was 58.9% in the 10 patients for whom the retention rate could be calculated using preoperative and postoperative magnetic resonance imaging (MRI). Postoperative MRI revealed small benign foci in two patients (16.7%), which were not palpable and did not become a clinical problem. A postoperative mammography revealed a small agglutinate calcification in one patient, which was determined to be benign through biopsy. CONCLUSIONS: Shaping the unaffected breast by autologous fat grafting combined with Brava is predictable, effective, and feasible as an aesthetic adjunct to unilateral breast reconstruction to achieve breast symmetry. LEVEL OF EVIDENCE: 4 Therapeutic.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4669992/

Record 1224

pmcid: PMC4537609

pmid: 26284186

doi:

title: Successful Excision of Gynecomastia with Nipple Repositioning Technique Utilizing the Dermoglandular Flap.

authors

- Motamed S
- Hassanpour SE
- Moosavizadeh SM
- Heidari A
- Rouientan A
- Nazemian M

journal: World journal of plastic surgery

journalAbbr: World J Plast Surg

year: 2015

pubdate: 2015 Jul

volume: 4

issue: 2

pages: 163-7

abstract: There are many surgical techniques for treating gynecomastia. We report a new surgical technique in an adolescent with fatty glandular gynecomastia grade III, who was referred from an endocrinologist to our clinic. We excised the gynecomastia with nipple repositioning utilizing the dermoglandular flap (about 1 cm thickness and 10 cm width). After one month, no complication was detected and the patient was satisfied with his new breasts. We suggest this technique for fatty glandular gynecomastia grade III.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4537609/

Record 1225

pmcid: PMC4569250

pmid: 26557823

doi: 10.1159/000381152

title: Surgical Strategies in the Treatment of Gynecomastia Grade I-II: The Combination of Liposuction and Subcutaneous Mastectomy Provides Excellent Patient Outcome and Satisfaction.

authors

- Schröder L
- Rudlowski C
- Walgenbach-Brünagel G
- Leutner C
- Kuhn W
- Walgenbach KJ

journal: Breast care (Basel, Switzerland)

journalAbbr: Breast Care (Basel)

year: 2015

pubdate: 2015 Jul

volume: 10

issue: 3

pages: 184-8

abstract: BACKGROUND: Gynecomastia (GM) is a benign condition with glandular tissue enlargement of the male breast. GM is classified into 4 grades of increasing severity. We describe a series of GM grade I-II, diagnosed, treated surgically and analyzed regarding feasibility, complication rate, and satisfaction. METHODS: From 2005 to 2012, a chart review was performed for 53 patients. Preoperative examination included endocrine and urological examination and exclusion of other pathological conditions. The surgical technique consisted of liposuction through an inframammarian-fold incision and excision of the glandular tissue by a minimal periareolar approach. RESULTS: A total number of 53 male patients with 104 breasts were available for analysis. By liposuction, a median of 300 ml (range: 10-1000 ml) was aspirated from each breast and 25.1 g (range: 3-233 g) gland tissue was resected. Surgery lasted between 25 and 164 min per patient (median: 72 min). 2 postoperative hemorrhages occurred (n = 2, 3.8%). 2 patients underwent re-operation due to cosmetic reasons (n = 2, 3.8%). CONCLUSIONS: This analysis demonstrates that treatment of GM grade I-II can easily be performed by liposuction combined with subcutaneous resection of the glandular tissue as a minimally invasive and low-impact surgical treatment with a low rate of complications and excellent patient satisfaction. Preoperative workup is important to rule out specific diseases or malignancy causing the GM.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4569250/

Record 1226

pmcid: PMC4372670

pmid: 25810917

doi: 10.1530/EDM-14-0122

title: Symptomatic empty sella syndrome: an unusual manifestation of Erdheim-Chester disease.

authors

- Loh WJ
- Sittampalam K
- Tan SC
- Chandran M

journal: Endocrinology, diabetes & metabolism case reports

journalAbbr: Endocrinol Diabetes Metab Case Rep

year: 2015

pubdate: 2015

volume: 2015

issue:

pages: 140122

abstract: UNLABELLED: Erdheim-Chester disease (ECD) is a potentially fatal condition characterized by infiltration of multiple organs by non-Langerhans histiocytes. Although endocrine dysfunction has been reported in association with ECD, to date, there have been no previous reports of empty sella syndrome (ESS) associated with it. We report the case of a patient with ECD who had symptomatic ESS. A 55-year-old man of Chinese ethnicity initially presented with symptoms of heart failure, fatigue and knee joint pain. Physical examination revealed xanthelasma, gynaecomastia, lung crepitations, hepatomegaly and diminished testicular volumes. He had laboratory evidence of hypogonadotrophic hypogonadism, secondary hypoadrenalism and GH deficiency. Imaging studies showed diffuse osteosclerosis of the long bones on X-ray, a mass in the right atrium and thickening of the pleura and of the thoracic aorta on fusion positron emission tomography-computed tomography. Magnetic resonance imaging (MRI) of the brain showed an empty sella. The diagnosis of ECD was confirmed by bone biopsy. LEARNING POINTS: ECD is a multisystemic disease that can affect the pituitary and other organs. The diagnosis of ECD is based on clinical and radiological features and histology, showing lipid-laden CD68(+) CD1a(-) S100(-) histiocytes surrounded by fibrosis.The finding of xanthelasmas especially in the presence of normal lipid levels in the presence of a multisystem infiltrative disorder should raise the suspicion of ECD.Systemic perturbation of autoimmunity may play a role in the pathogenesis of ECD and is an area that merits further research.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4372670/

Record 1227

pmcid: PMC4438533

pmid: 25928084

doi: 10.1186/s12967-015-0492-2

title: Systems biology analysis reveals NFAT5 as a novel biomarker and master regulator of inflammatory breast cancer.

authors

- Remo A
- Simeone I
- Pancione M
- Parcesepe P
- Finetti P
- Cerulo L
- Bensmail H
- Birnbaum D
- Van Laere SJ
- Colantuoni V
- Bonetti F
- Bertucci F
- Manfrin E

- Ceccarelli M

journal: Journal of translational medicine

journalAbbr: J Transl Med

year: 2015

pubdate: 2015 May 1

volume: 13

issue:

pages: 138

abstract: BACKGROUND: Inflammatory breast cancer (IBC) is the most rare and aggressive variant of breast cancer (BC); however, only a limited number of specific gene signatures with low generalization abilities are available and few reliable biomarkers are helpful to improve IBC classification into a molecularly distinct phenotype. We applied a network-based strategy to gain insight into master regulators (MRs) linked to IBC pathogenesis. METHODS: In-silico modeling and Algorithm for the Reconstruction of Accurate Cellular Networks (ARACNe) on IBC/non-IBC (nIBC) gene expression data (n = 197) was employed to identify novel master regulators connected to the IBC phenotype. Pathway enrichment analysis was used to characterize predicted targets of candidate genes. The expression pattern of the most significant MRs was then evaluated by immunohistochemistry (IHC) in two independent cohorts of IBCs (n = 39) and nIBCs (n = 82) and normal breast tissues (n = 15) spotted on tissue microarrays. The staining pattern of non-neoplastic mammary epithelial cells was used as a normal control. RESULTS: Using in-silico modeling of network-based strategy, we identified three top enriched MRs (NFAT5, CTNNB1 or β-catenin, and MGA) strongly linked to the IBC phenotype. By IHC assays, we found that IBC patients displayed a higher number of NFAT5-positive cases than nIBC (69.2% vs. 19.5%; p-value = 2.79 10(-7)). Accordingly, the majority of NFAT5-positive IBC samples revealed an aberrant nuclear expression in comparison with nIBC samples (70% vs. 12.5%; p-value = 0.000797). NFAT5 nuclear accumulation occurs regardless of WNT/β-catenin activated signaling in a substantial portion of IBCs, suggesting that NFAT5 pathway activation may have a relevant role in IBC pathogenesis. Accordingly, cytoplasmic NFAT5 and membranous β-catenin expression were preferentially linked to nIBC, accounting for the better prognosis of this phenotype. CONCLUSIONS: We provide evidence that NFAT-signaling pathway activation could help to identify aggressive forms of BC and potentially be a guide to assignment of phenotype-specific therapeutic agents. The NFAT5 transcription factor might be developed into routine clinical practice as a putative biomarker of IBC phenotype.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4438533/

Record 1228

pmcid: PMC10852633

pmid: 25392187

doi: 10.1177/1533317514556874

title: Testosterone treatment of men with mild cognitive impairment and low testosterone levels.

authors

- Cherrier MM
- Anderson K
- Shofer J
- Millard S
- Matsumoto AM

journal: American journal of Alzheimer's disease and other dementias

journalAbbr: Am J Alzheimers Dis Other Demen

year: 2015

pubdate: 2015 Jun

volume: 30

issue: 4

pages: 421-30

abstract: INTRODUCTION: This study investigated the effects of testosterone (T) treatment on cognition, mood, and quality of life in men with mild cognitive impairment (MCI) and low serum T levels. METHODS: A total of 351 community-dwelling men were screened, and 37 men evidenced both MCI and low T of whom 27 agreed for further screening. Twenty-two met all the study inclusion/exclusion criteria and enrolled in a 6-month randomized, double-blind, placebo-controlled study. RESULTS: Total T levels significantly increased in the T treatment group. No significant changes were observed in measures of cognition, mood, or quality of life other than improvement in 1 objective measure of verbal memory (P < .05) and decreased depression symptoms (P < .02) in the treatment group. CONCLUSIONS: Testosterone treatment may modestly improve verbal memory and depression symptoms in men with both MCI and low T.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10852633/

Record 1229

pmcid: PMC4325408

pmid: 25709705

doi:

title: The effect of aromatherapy massage with lavender oil on severity of primary dysmenorrhea in Arsanjan students.

authors

- Bakhtshirin F
- Abedi S
- YusefiZoj P
- Razmjooee D

journal: Iranian journal of nursing and midwifery research

journalAbbr: Iran J Nurs Midwifery Res

year: 2015

pubdate: 2015 Jan-Feb

volume: 20

issue: 1

pages: 156-60

abstract: BACKGROUND: Presently, using complementary therapy such as lavender oil has specific application in medicine. The purpose of this study was to investigate the effect of aromatherapy massage on the severity of primary dysmenorrhea in nursing and midwifery students of Islamic Azad University of Arsanjan, Iran. MATERIALS AND METHODS: This study was performed using clinical trial method on 80 eligible students whose level of pain was measured by visual analogue scale (VAS) before the intervention. Each participant, in the first days of menstruation, randomly received two types of massage with lavender and placebo oil in two consecutive cycles of menstruation. Their level of pain was measured before and 30 min after the intervention. In this study, each group was considered as their self-control group in the next cycle. The data were analyzed by SPSS software. RESULTS: A significant decrease in VAS score after lavender massage was detected in comparison with placebo massage. There was a statistically significant difference between VAS scores after and before placebo massage. In addition, statistically the effect of lavender massage on the severity of primary dysmenorrhea was higher than that of placebo massage (P < 0.001). CONCLUSIONS: Findings of this study showed that lavender oil massage decreases primary dysmenorrhea and it can be used as an effective herbal

drug.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4325408/

Record 1230

pmcid: PMC4579177

pmid: 26430637

doi: 10.5999/aps.2015.42.5.630

title: Treatment of Pseudoangiomatous Stromal Hyperplasia of the Breast: Implant-Based Reconstruction with a Vascularized Dermal Sling.

authors

- Jung BK
- Nahm JH
- Lew DH
- Lee DW

journal: Archives of plastic surgery

journalAbbr: Arch Plast Surg

year: 2015

pubdate: 2015 Sep

volume: 42

issue: 5

pages: 630-4

abstract: Pseudoangiomatous stromal hyperplasia (PASH) of the breast is a benign mesenchymal lesion with incidental histologic findings. Surgical excision is recommended as the treatment of choice for PASH, although the recurrence rates after excision range from 15% to 22%. A 46-year-old-female presented with a six-month history of bilateral breast enlargement and painful sensation mimicking inflammatory carcinoma. Imaging studies demonstrated innumerable enhancing nodules in both breasts. Due to the growth of the lesions and progressive clinical symptoms, bilateral subcutaneous mastectomy was performed. Grossly, the specimens were round and well-circumscribed, and the histologic examination revealed PASH. After mastectomy, we created a pocket with the pectoralis major muscle and a lower skin flap, which was deepithelized. Anatomical mammary implants were inserted, and the nipple areolar complex was transferred to a new position as a free graft. The aesthetic result was satisfactory after twelve months of follow-up.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4579177/

Record 1231

pmcid: PMC4670967

pmid: 26668786

doi: 10.3205/iprs000070

title: Unilateral gynaecomastia in a 16-month-old boy with neurofibromatosis type 1 - case report and brief review of the literature.

authors

- Friedrich RE
- Hagel C
- Mautner VF

journal: GMS Interdisciplinary plastic and reconstructive surgery DGPW

journalAbbr: GMS Interdiscip Plast Reconstr Surg DGPW

year: 2015

pubdate: 2015

volume: 4

issue:

pages: Doc11

abstract: Neurofibromatosis type 1 (NF1) is an autosomal dominant disease that shows high penetrance with a wide variability in the phenotype. Prepubertal enlargement of the breast in male subjects affected by this condition is well known, but rarely reported. The present case report describes diagnosis and therapy of unilateral gynaecomastia in a toddler showing integumental stigmata of NF1. Furthermore, the report provides a brief review of the literature concerning this finding in NF1. According to this review, the present case appears to be one of the youngest NF1-affected males affected by gynaecomastia that has been reported.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4670967/

Record 1232

pmcid: PMC8627700

pmid: 26487173

doi: 10.1002/14651858.CD007258.pub3

title: Wound drainage after plastic and reconstructive surgery of the breast.

authors

- Khan SM
- Smeulders MJ
- Van der Horst CM

journal: The Cochrane database of systematic reviews

journalAbbr: Cochrane Database Syst Rev

year: 2015

pubdate: 2015 Oct 21

volume: 2015

issue: 10

pages: CD007258

abstract: BACKGROUND: Wound drains are often used after plastic and reconstructive surgery of the breast, in order to reduce potential complications. It is unclear whether there is any evidence to support this practice and we therefore undertook a systematic review of the best evidence available. OBJECTIVES: To compare the safety and efficacy of the use of wound drains following elective plastic and reconstructive surgery procedures of the breast. SEARCH METHODS: For the first update of this review we searched the Cochrane Wounds Group Specialised Register (searched 4 March 2015); The Cochrane Central Register of Controlled Trials (CENTRAL) (The Cochrane Library 2015, Issue 2); Ovid MEDLINE (2012 to March 3 2015); Ovid MEDLINE (In-Process & Other Non-Indexed Citations March 3 2015); Ovid EMBASE (2012 to March 3 2015); and EBSCO CINAHL (2012 to March 4 2015). There were no restrictions on the basis of date or language of publication. SELECTION CRITERIA: Three review authors undertook independent screening of the search results. All randomised trials (RCTs) that compared the use of a wound drain with no wound drain following plastic and reconstructive surgery of the breast (breast augmentation, breast reduction and breast reconstruction) in women were eligible. DATA COLLECTION AND ANALYSIS: Two review authors undertook independent data extraction of study characteristics, methodological quality and outcomes (e.g. infection, other wound complications, pain, and length of hospital stay). Risk of bias was assessed independently by two review authors. We calculated the risk ratio (RR) for dichotomous outcomes and mean differences (MD) for continuous outcomes, with 95%

confidence intervals. Analysis was on an intention-to-treat basis. MAIN RESULTS: Three randomised trials were identified and included in the review out of 190 studies that were initially screened; all evaluated wound drainage after breast reduction surgery. No new trials were identified for this first update. In total there were 306 women in the three trials, and 505 breasts were studied (254 drained, and 251 who were not drained). Apart from a significantly shorter duration of hospital stay for those participants who did not have drains (MD 0.77; 95% CI 0.40 to 1.14), there was no statistically significant impact of the use of drains on outcomes. AUTHORS' CONCLUSIONS: The limited evidence available shows no significant benefit of using post-operative wound drains in reduction mammoplasty, though hospital stay may be shorter when drains are not used. No data are available for breast augmentation or breast reconstruction, and this requires investigation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8627700/

Record 1233

pmcid: PMC4177599

pmid: 25240484

doi: 10.1186/1472-6963-14-413

title: 'Cosmetic boob jobs' or evidence-based breast surgery: an interpretive policy analysis of the rationing of 'low value' treatments in the English National Health Service.

authors

- Russell J
- Swinglehurst D
- Greenhalgh T

journal: BMC health services research

journalAbbr: BMC Health Serv Res

year: 2014

pubdate: 2014 Sep 20

volume: 14

issue:

pages: 413

abstract: BACKGROUND: In England the National Health Service (NHS) is not allowed to impose 'blanket bans' on treatments, but local commissioners produce lists of 'low value' procedures that they will normally not fund. Breast surgery is one example. However, evidence suggests that some breast surgery is clinically effective, with significant health gain. National guidelines indicate the circumstances under which breast surgery should be made available on the NHS, but there is widespread variation in their implementation.The purpose of this study was to explore the work practices of 'individual funding request' (IFR) panels, as they considered 'one-off' funding requests for breast surgery; examine how the notion of 'value' is dialogically constructed, and how decisions about who is deserving of NHS funding and who is not are accomplished in practice. METHODS: We undertook ethnographic exploration of three IFR panels. We extracted all (22) breast surgery cases considered by these panels from our data set and progressively focused on three case discussions, one from each panel, covering the three main breast procedures.We undertook a microanalysis of the talk and texts arising from these cases, within a conceptual framework of interpretive policy analysis. RESULTS: Through an exploration of the symbolic artefacts (language, objects and acts) that are significant carriers of policy meaning, we identified the ways in which IFR panels create their own 'interpretive communities', within which deliberations about the funding of breast surgery are differently framed, and local decisions come to be justified. In particular, we demonstrated how each decision was contingent on [a] the evaluative accent given to certain words, [b] the work that documentary objects achieve in foregrounding particular concerns, and [c] the act of categorising. Meaning was constructed dialogically through local interaction and broader socio-cultural discourses about breasts and 'cosmetic' surgery. CONCLUSION: Despite the appeal of calls to tackle 'unwarranted variation' in access to low priority treatments by ensuring uniformity of local guidelines and policies, our findings suggest that ultimately, given the contingent nature of practice, this is likely to remain an illusory policy goal. Our findings

challenge the scientistic thinking underpinning mainstream health policy discourse.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4177599/

Record 1234

pmcid: PMC4113269

pmid: 25077171

doi: 10.1002/mgg3.76

title: 46,XY disorder of sexual development resulting from a novel monoallelic mutation (p.Ser31Phe) in the steroid 5α-reductase type-2 (SRD5A2) gene.

authors

- Chávez B
- Ramos L
- Gómez R
- Vilchis F

journal: Molecular genetics & genomic medicine

journalAbbr: Mol Genet Genomic Med

year: 2014

pubdate: 2014 Jul

volume: 2

issue: 4

pages: 292-6

abstract: Inactivating mutations of the 5α-steroid reductase type-2 (SRD5A2) gene result in a broad spectrum of masculinization defects, ranging from a male phenotype with hypospadias to a female phenotype with Wolffian structures. Molecular studies of the SRD5A2 revealed a new heterozygous gene variant within the coding region that results in phenotypic expression. A c.92C>T transition changing serine to phenylalanine at codon 31 of exon 1 (p.Ser31Phe) was identified in a patient with 46,XY disorder of sexual development who displayed glandular hypospadias with micropenis and bilateral cryptorchidism. The restoration of the p.Ser31Phe mutation by site-directed mutagenesis and transient expression assays using cultured HEK-293 cells showed that this novel substitution does not abolish but does deregulate the catalytic efficiency of the enzyme. Thus, the maximum velocity (V max) value was higher for the mutant enzyme (22.5 ± 6.9 nmol DHT mg protein(-1) h(-1)) than for the wild-type enzyme (9.8 ± 2.0 nmol DHT mg protein(-1) h(-1)). Increased in vitro activity of the p.Ser31Phe mutant suggested an activating effect. This case provides evidence that heterozygous missense mutations in SRD5A2 may induce the abnormal development of male external genitalia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4113269/

Record 1235

pmcid: PMC3903163

pmid: 24456024

doi: 10.1089/bfm.2014.9996

title: ABM clinical protocol #2: Guidelines for hospital discharge of the breastfeeding term newborn and mother: "The going home protocol," revised 2014.

authors

- Evans A
- Marinelli KA

- Taylor JS

journal: Breastfeeding medicine : the official journal of the Academy of Breastfeeding Medicine

journalAbbr: Breastfeed Med

year: 2014

pubdate: 2014 Jan-Feb

volume: 9

issue: 1

pages: 3-8

abstract: A central goal of The Academy of Breastfeeding Medicine is the development of clinical protocols for managing common medical problems that may impact breastfeeding success. These protocols serve only as guidelines for the care of breastfeeding mothers and infants and do not delineate an exclusive course of treatment or serve as standards of medical care. Variations in treatment may be appropriate according to the needs of an individual patient.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3903163/

Record 1236

pmcid: PMC4571518

pmid: 26396480

doi: 10.1007/s12262-013-0887-3

title: An Ameliorated Approach for Sharp Resection in Gynecomastia Surgery.

authors

- Hoşnuter M

journal: The Indian journal of surgery

journalAbbr: Indian J Surg

year: 2014

pubdate: 2014 Oct

volume: 76

issue: 5

pages: 419-24

abstract: Surgery has become the accepted standard for the majority of patients with gynecomastia to get rid of feminine-looking breast enlargement. Many surgical techniques have been proposed according to grade of gynecomastia. The sharp resection of glandular tissues is a keystone for most of them. However, technical difficulties in application and inexperience in mastectomy often lead to poor cosmetic outcomes. Over excision and saucer-like deformity, under resection, and asymmetries are most common ones among them. The author presents an ameliorated subcutaneous mastectomy method to facilitate the learning curve and to improve the esthetic results avoiding saucer-like deformity and other breast contour deformities. This method consists of an en bloc tissue dissection via superior periareolar incision and excision of fatty glandular tissue in suprafascial plan, with slicing and trimming procedure. It is possible to adjust the degree of tissue reduction during surgery; hence, it may be labeled as a "cut-as-you-go" technique. Between 2008 and 2012, 23 male patients were operated with this technique. Medical photographs and drawings were used to describe the technique. The operation resulted in smooth, symmetric breasts befitting to men in all 23 patients. No major complications were observed in any of the cases. None of the patients reported a discomfort in sensation of nipple-areolar complex. The presented technique provides high degree of patient satisfaction and excellent esthetic outcomes and is a promising choice in gynecomastia surgery with extremely low recurrence rates and easy learning curve.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4571518/

Record 1237

pmcid: PMC4168024

pmid: 24758365

doi: 10.1517/14656566.2014.913022

title: An update on male hypogonadism therapy.

authors

- Surampudi P
- Swerdloff RS
- Wang C

journal: Expert opinion on pharmacotherapy

journalAbbr: Expert Opin Pharmacother

year: 2014

pubdate: 2014 Jun

volume: 15

issue: 9

pages: 1247-64

abstract: INTRODUCTION: Men who have symptoms associated with persistently low serum total testosterone level should be assessed for testosterone replacement therapy. AREAS COVERED: Acute and chronic illnesses are associated with low serum testosterone and these should be recognized and treated. Once the diagnosis of male hypogonadism is made, the benefits of testosterone treatment usually outweigh the risks. Without contraindications, the patient should be offered testosterone replacement therapy. The options of testosterone delivery systems (injections, transdermal patches/gels, buccal tablets, capsules and implants) have increased in the last decade. Testosterone improves symptoms and signs of hypogonadism such as sexual function and energy, increases bone density and lean mass and decreases visceral adiposity. In men who desire fertility and who have secondary hypogonadism, testosterone can be withdrawn and the patients can be placed on gonadotropins. New modified designer androgens and selective androgen receptor modulators have been in preclinical and clinical trials for some time. None of these have been assessed for the treatment of male hypogonadism. EXPERT OPINION: Despite the lack of prospective long-term data from randomized, controlled clinical trials of testosterone treatment on prostate health and cardiovascular disease risk, the available evidence suggests that testosterone therapy should be offered to symptomatic hypogonadal men.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4168024/

Record 1238

pmcid: PMC4094624

pmid: 25045284

doi: 10.2147/PROM.S52788

title: Androgen deprivation therapy for prostate cancer: long-term safety and patient outcomes.

authors

- Ahmadi H
- Daneshmand S

journal: Patient related outcome measures

journalAbbr: Patient Relat Outcome Meas

year: 2014

pubdate: 2014

volume: 5

issue:

pages: 63-70

abstract: Androgen deprivation therapy (ADT) constitutes the first-line treatment for patients with locally advanced tumors, recurrent or metastatic disease. Given its widespread use, clinicians should be familiar with common side effects of this treatment. This review focuses on common side effects of ADT and available treatment options to control the side effects. Also, it briefly compares continuous ADT with other therapeutic approaches for androgen deprivation in prostate cancer patients. Similar to hormonal medications, newer non-hormonal therapeutic options including gabapentin and acupuncture have at best moderate effect in controlling hot flashes in patients on ADT. Supervised and/or home exercise programs significantly improve ADT-related fatigue, metabolic/cardiovascular side effects, and cognitive dysfunction. Denosumab, a human monoclonal antibody against RANK-L, is more effective than bisphosphonates in preventing skeletal-related events in patients with metastatic or castrate-resistant prostate cancer and unlike bisphosphonates, it can also reduce the risk of vertebral fractures in men receiving ADT for non-metastatic prostate cancer. Toremifene, a selective estrogen receptor inhibitor, has dual beneficial effects on ADT-related osteoporosis and metabolic dysfunction. Metformin coupled with lifestyle modification is also a well-tolerated treatment for metabolic changes during ADT. While producing similar oncological outcomes, intermittent ADT is associated with higher quality of life in patients under ADT by improving bone health, less metabolic and hematologic complications, and fewer hot flashes and sexual dysfunction events.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4094624/

Record 1239

pmcid: PMC3975166

pmid: 24552677

doi: 10.1093/jnci/djt465

title: Anthropometric and hormonal risk factors for male breast cancer: male breast cancer pooling project results.

authors

- Brinton LA
- Cook MB
- McCormack V
- Johnson KC
- Olsson H
- Casagrande JT
- Cooke R
- Falk RT
- Gapstur SM
- Gaudet MM
- Gaziano JM
- Gkiokas G
- Guénel P
- Henderson BE
- Hollenbeck A
- Hsing AW
- Kolonel LN

- Isaacs C
- Lubin JH
- Michels KB
- Negri E
- Parisi D
- Petridou ET
- Pike MC
- Riboli E
- Sesso HD
- Snyder K
- Swerdlow AJ
- Trichopoulos D
- Ursin G
- van den Brandt PA
- Van Den Eeden SK
- Weiderpass E
- Willett WC
- Ewertz M
- Thomas DB

journal: Journal of the National Cancer Institute

journalAbbr: J Natl Cancer Inst

year: 2014

pubdate: 2014 Mar

volume: 106

issue: 3

pages: djt465

abstract: BACKGROUND: The etiology of male breast cancer is poorly understood, partly because of its relative rarity. Although genetic factors are involved, less is known regarding the role of anthropometric and hormonally related risk factors. METHODS: In the Male Breast Cancer Pooling Project, a consortium of 11 case-control and 10 cohort investigations involving 2405 case patients (n = 1190 from case-control and n = 1215 from cohort studies) and 52013 control subjects, individual participant data were harmonized and pooled. Unconditional logistic regression generated study design-specific (case-control/cohort) odds ratios (ORs) and 95% confidence intervals (CIs), with exposure estimates combined using fixed effects meta-analysis. All statistical tests were two-sided. RESULTS: Risk was statistically significantly associated with weight (highest/lowest tertile: OR = 1.36; 95% CI = 1.18 to 1.57), height (OR = 1.18; 95% CI = 1.01 to 1.38), and body mass index (BMI; OR = 1.30; 95% CI = 1.12 to 1.51), with evidence that recent rather than distant BMI was the strongest predictor. Klinefelter syndrome (OR = 24.7; 95% CI = 8.94 to 68.4) and gynecomastia (OR = 9.78; 95% CI = 7.52 to 12.7) were also statistically significantly associated with risk, relations that were independent of BMI. Diabetes also emerged as an independent risk factor (OR = 1.19; 95% CI = 1.04 to 1.37). There were also suggestive relations with cryptorchidism (OR = 2.18; 95% CI = 0.96 to 4.94) and orchitis (OR = 1.43; 95% CI = 1.02 to 1.99). Although age at onset of puberty and histories of infertility were unrelated to risk, never having had children was statistically significantly related (OR = 1.29; 95% CI = 1.01 to 1.66). Among individuals diagnosed at older ages, a history of fractures was statistically significantly related (OR = 1.41; 95% CI = 1.07 to 1.86). CONCLUSIONS: Consistent findings across case-control and cohort investigations, complemented by pooled analyses, indicated important roles for anthropometric and hormonal risk factors in the etiology of male breast cancer. Further investigation should focus on potential roles of endogenous hormones.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3975166/

Record 1240

pmcid: PMC4189794

pmid: 25296284

doi: 10.1371/journal.pone.0109039

title: Association between maternal exposure to di(2-ethylhexyl) phthalate and reproductive hormone levels in fetal blood: the Hokkaido study on environment and children's health.

authors

- Araki A
- Mitsui T
- Miyashita C
- Nakajima T
- Naito H
- Ito S
- Sasaki S
- Cho K
- Ikeno T
- Nonomura K
- Kishi R

journal: PloS one

journalAbbr: PLoS One

year: 2014

pubdate: 2014

volume: 9

issue: 10

pages: e109039

abstract: Prenatal di(2-ethylhexyl) phthalate (DEHP) exposure can produce reproductive toxicity in animal models. Only limited data exist from human studies on maternal DEHP exposure and its effects on infants. We aimed to examine the associations between DEHP exposure in utero and reproductive hormone levels in cord blood. Between 2002 and 2005, 514 pregnant women agreed to participate in the Hokkaido Study Sapporo Cohort. Maternal blood samples were taken from 23-35 weeks of gestation and the concentration of the primary metabolite of DEHP, mono(2-ethylhexyl) phthalate (MEHP), was measured. Concentrations of infant reproductive hormones including estradiol (E2), total testosterone (T), and progesterone (P4), inhibin B, insulin-like factor 3 (INSL3), steroid hormone binding globulin, follicle-stimulating hormone, and luteinizing hormone were measured from cord blood. Two hundred and two samples with both MEHP and hormones' data were included in statistical analysis. The participants completed a self-administered questionnaire regarding information on maternal characteristics. Gestational age, birth weight and infant sex were obtained from birth records. In an adjusted linear regression analysis fit to all study participants, maternal MEHP levels were found to be associated with reduced levels of T/E2, P4, and inhibin B. For the stratified analyses for sex, inverse associations between maternal MEHP levels T/E2, P4, inhibin B, and INSL3 were statistically significant for males only. In addition, the MEHP quartile model showed a significant p-value trend for P4, inhibin B, and INSL3 decrease in males. Since inhibin B and INSL3 are major secretory products of Sertoli and Leydig cell, respectively, the results of this study suggest that DEHP exposure in utero may have adverse effects on both Sertoli and Leydig cell development in males, which agrees with the results obtained from animal studies. Comprehensive studies investigating phthalates' exposure in humans, as well as their long-term effects on reproductive development are needed.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4189794/

Record 1241

pmcid: PMC4061513

pmid: 24886365

doi: 10.1186/1756-9966-33-47

title: AZD8931, an equipotent, reversible inhibitor of signaling by epidermal growth factor receptor (EGFR), HER2, and HER3: preclinical activity in HER2 non-amplified inflammatory breast cancer models.

authors

- Mu Z
- Klinowska T
- Dong X
- Foster E
- Womack C
- Fernandez SV
- Cristofanilli M

journal: Journal of experimental & clinical cancer research : CR

journalAbbr: J Exp Clin Cancer Res

year: 2014

pubdate: 2014 May 30

volume: 33

issue: 1

pages: 47

abstract: INTRODUCTION: Epidermal growth factor receptor (EGFR) overexpression has been associated with prognostic and predictive value in inflammatory breast cancer (IBC). Epidermal growth factor receptor 2 (HER2) overexpression is observed at a higher rate in IBC compared with noninflammatory breast cancer. Current clinically available anti-HER2 therapies are effective only in patients with HER2 amplified breast cancer, including IBC. AZD8931 is a novel small-molecule equipotent inhibitor of EGFR, HER2, and HER3 signaling. In this study, we investigated the antitumor activity of AZD8931 alone or in combination with paclitaxel using preclinical models of EGFR-overexpressed and HER2 non-amplified IBC cells. METHODS: Two IBC cell lines SUM149 and FC-IBC-02 derived from pleural effusion of an IBC patient were used in this study. Cell growth and apoptotic cell death were examined in vitro. For the in vivo tumor growth studies, IBC cells were orthotopically transplanted into the mammary fat pads of immunodeficient mice. AZD8931 was given by daily oral gavage at doses of 25 mg/kg, 5 days/week for 4 weeks. Paclitaxel was subcutaneously injected twice weekly. RESULTS: AZD8931 significantly suppressed cell growth of IBC cells and induced apoptosis of human IBC cells in vitro. Significantly, we showed that AZD8931 monotherapy inhibited xenograft growth and the combination of paclitaxel + AZD8931 was demonstrably more effective than paclitaxel or AZD8931 alone treatment at delaying tumor growth in vivo in orthotopic IBC models. CONCLUSION: AZD8931 single agent and in combination with paclitaxel demonstrated signal inhibition and antitumor activity in EGFR-overexpressed and HER2 non-amplified IBC models. These results suggest that AZD8931 may provide a novel therapeutic strategy for the treatment of IBC patients with HER2 non-amplified tumors.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4061513/

Record 1242

pmcid: PMC4269278

pmid: 25544925

doi: 10.1155/2014/250608

title: Bilateral diffuse tumorous pseudoangiomatous stromal hyperplasia: a case of bilateral mastectomy in a 29-year-old woman.

authors

- Dai H
- Connor C
- Cui W
- Gatewood J
- Fan F

journal: Case reports in pathology

journalAbbr: Case Rep Pathol

year: 2014

pubdate: 2014

volume: 2014

issue:

pages: 250608

abstract: Pseudoangiomatous stromal hyperplasia (PASH) is a benign breast lesion commonly encountered as an incidental microscopic finding. However, it can also manifest as a mass-forming lesion (tumorous PASH) capable of recurrence after surgical excision. Most of the previously reported cases of tumorous PASH present as a single dominant mass. Here we reported a rare case of diffuse tumorous PASH involving bilateral breasts clinically mimicking malignancy. A 29-year-old African-American female presented with a one-year history of bilateral breast enlargement and asymmetry. Physical examination revealed multiple palpable nodules in bilateral breasts. Imaging studies demonstrated innumerable homogeneously enhancing masses throughout both breasts, greater on the left, with multiple cysts and edema. Biopsy of the breast nodules demonstrated histopathological changes consistent with PASH. Due to the extent of the lesions and progressive clinical symptoms, decision was made to perform bilateral mastectomy. Macroscopic examination of the bilateral mastectomy specimens revealed markedly enlarged breasts with marked edema and numerous well-defined firm nodules. Microscopic evaluation of the nodules confirmed the diagnosis of PASH. No evidence of malignancy was identified. Recognition of this rare form of PASH is essential for the proper clinical management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4269278/

Record 1243

pmcid: PMC4069713

pmid: 24957586

doi: 10.1136/bcr-2014-204535

title: Breast augmentation with an unknown substance.

authors

- Ebrahim L
- Morrison D
- Kop A
- Taylor D

journal: BMJ case reports

journalAbbr: BMJ Case Rep

year: 2014

pubdate: 2014 Jun 23

volume: 2014

issue:

pages:

abstract: Before the widespread use of silicone implants various foreign substances were injected directly into the breasts. The nature of these materials sometimes remains unknown and can cause various complications requiring surgical intervention. Preoperative diagnostic imaging can help characterise the type and distribution of the injected material, thereby assisting in making decisions regarding treatment. We report a case of breast augmentation with an unknown substance, aiming to highlight some imaging characteristics of different breast augmentation substances.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4069713/

Record 1244

pmcid: PMC4263805

pmid: 25099481

doi: 10.1007/s13244-014-0344-2

title: Breast imaging findings in haematological malignancies.

authors

- Glazebrook KN
- Zingula S
- Jones KN
- Fazzio RT

journal: Insights into imaging

journalAbbr: Insights Imaging

year: 2014

pubdate: 2014 Dec

volume: 5

issue: 6

pages: 715-22

abstract: OBJECTIVES: The objectives of this article are to review and illustrate the imaging appearances of haematological malignancies in the breast. METHODS: With Institutional Review Board approval, a search of the surgical pathology records from 1st January 2000 to 1st July 2012 was performed for haematological malignancies. RESULTS: Forty-eight cases of haematological malignancies (42 women and 6 men) were identified with imaging available for review: 39 cases of breast lymphoma, 6 cases of chronic lymphocytic leukaemia, 2 cases of acute leukaemia and 1 case of known multiple myeloma. CONCLUSIONS: Breast manifestations of haematological malignancies are rare. They can have a variable appearance at imaging and can mimic primary breast carcinoma. In the setting of suspicious breast imaging findings, pathological diagnosis of haematological malignancy is concordant. Correlation with a clinical history of prior haematological malignancy can be helpful in suggesting the diagnosis and help prevent unnecessary surgical treatment. TEACHING POINTS: • Breast haematological malignancies are rare but the imaging appearances can mimic breast carcinoma. • Breast lymphoma, most often B-cell non-Hodgkin lymphoma, may be primary or due to secondary disease. • At ultrasound, haematological malignancies may present as a heterogeneous or predominantly echogenic mass. • Haematological malignancies show intense activity on PET/CT except myeloma which has low FDG uptake.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4263805/

Record 1245

pmcid: PMC3900015

pmid: 24761340

doi: 10.1089/wound.2012.0404

title: Breast Reconstruction After Radiotherapy.

authors

- Yoshimoto H
- Hamuy R

journal: Advances in wound care

journalAbbr: Adv Wound Care (New Rochelle)

year: 2014

pubdate: 2014 Jan 1

volume: 3

issue: 1

pages: 12-15

abstract: It was once common practice in the treatment of breast cancer for total mastectomy and axillary lymph node dissection to be conventionally performed. However, breast-conserving surgery is increasingly being performed with marked improvement in a multidisciplinary treatment approach, including surgery, chemotherapy, irradiation, and antihormonal therapy. We must choose the optimal reconstructive methods with the reduction of the excision range. Furthermore, we also consider the local chronic radiation injury after adjuvant radiotherapy on breast reconstruction. As for breast reconstruction, the use of autologous tissues or artificial prostheses is common. However, after radiotherapy or if radiotherapy is planned, the complications such as infection, skin necrosis, or exposure of the implant are increased in breast reconstruction with implants. Therefore, the breast is reconstructed with autologous tissue mainly with radiotherapy using an autologous flap transfer. Meanwhile, the autologous fat transfer with adipose-derived regenerative cells for repair and regeneration has recently been investigated in reconstructive surgery. We discuss the autologous flap and fat transfer for breast reconstruction.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3900015/

Record 1246

pmcid: PMC4718330

pmid: 25916019

doi:

title: Breast self examination and mammography in cancer screening: women health protective behavior.

authors

- Ghodsi Z
- Hojjatoleslami S

journal: Journal of preventive medicine and hygiene

journalAbbr: J Prev Med Hyg

year: 2014

pubdate: 2014 Jun

volume: 55

issue: 2

pages: 46-9

abstract: BACKGROUND: Breast cancer (BC) is one of the leading causes of death among women. Secondary prevention may enable early detection, but this is suboptimal among all Iranian women. METHODS: This was a descriptive, analytic cross sectional study on 385 women 35 years old or more with no history of BC. Participants were selected by simple randomized method and were assessed through a two-part self-administered questionnaire and a self-examination checklist with content validity and test-re-test reliability. RESULTS: 14.8% of women carried out breast self examination (BSE). Among them 5.7% was done in adequate timing and 9.4% performed it on a regular basis. The average age of BSE onset was 20.1 ± 7.6 and mean of Score was 6.25 ± 2.26 (2-11). 2.3% of participants performed BSE poorly, 7.5% fairly and 1.6% performed it well. 25.84% of samples had a history of mammography that 13% of whom received it as a result of prescription. The average age for mammography was 36 ± 7.2 (20-50) years and the frequency of mammography was 1.8 ± 1.4 (1-8) of times. Due to the low percentage of breast cancer preventive behaviors, in this study knowledge towards breast cancer was also measured because they are factors that are crucial in performance. CONCLUSION: The results highlight the need to educate Iranian women to recognize the risk factors to promote early detection of breast cancer. Creation of health behavioral by focused educational programs might cause decrease of breast cancer prevalence.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4718330/

Record 1247

pmcid: PMC4173862

pmid:

doi:

title: Canadian Society for Aesthetic Plastic Surgery 41st Annual Meeting: September 19 & 20, 2014 Toronto, Ontario.

authors

- Pritchard S
- Carr N

journal: Plastic Surgery

journalAbbr: Plast Surg (Oakv)

year: 2014

pubdate: 2014 Autumn

volume: 22

issue: 3

pages: 168-70

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4173862/

Record 1248

pmcid: PMC4013055

pmid: 24773629

doi: 10.1186/1472-6823-14-38

title: Central precocious puberty in a 3 year-old girl with Phenylketonuria: a rare association?

authors

- Lucaccioni L
- Schwahn BC

- Donaldson M
- Giacomozzi C

journal: BMC endocrine disorders

journalAbbr: BMC Endocr Disord

year: 2014

pubdate: 2014 Apr 28

volume: 14

issue:

pages: 38

abstract: BACKGROUND: Central precocious puberty (CPP) and phenylketonuria (PKU) are two rare conditions, the latter being the rarer. To date, only one case featuring both these conditions has been reported, and hyperphenylalaninemia was assumed triggering CPP. CASE PRESENTATION: We present a 3.2 years old girl referred with a 12 months history of breast and pubic hair development, and vaginal discharge. Hyperphenylalaninemia had been identified by newborn screening and PKU subsequently confirmed by plasma amino acid and genetic analysis. Early dietary control of plasma phenylalanine had been excellent afterwards, resulting in phenylalanine concentrations consistently within the recommended range. Clinical scenario, hormonal assessment and imaging were in keeping with true idiopathic central precocious puberty. Treatment with long lasting gonadotropin-releasing hormone analogue led to regression of secondary sexual characteristics. CONCLUSION: We describe for the first time CPP in a girl affected with PKU but with persistently well controlled blood phenylalanine concentrations. This finding is in contrast to a previous report which suggested persistently high phenylalaninemia levels as potential trigger for CPP in PKU patients. Our report, together with the lack of evidence in published cohort studies of children with PKU, strongly suggests this rare association is coincidental and independent of the presence of severe hyperphenylalaninemia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4013055/

Record 1249

pmcid: PMC4275969

pmid: 25437663

doi: 10.1016/j.ijscr.2014.11.040

title: Clinical and surgical management of unilateral prepubertal gynecomastia.

authors

- Ferraro GA
- De Francesco F
- Romano T
- Grandone A
- D'Andrea F
- Miraglia Del Giudice E
- Perrone L
- Nicoletti G

journal: International journal of surgery case reports

journalAbbr: Int J Surg Case Rep

year: 2014

pubdate: 2014

volume: 5

issue: 12

pages: 1158-61

abstract: INTRODUCTION: Gynecomastia is the benign proliferation of the glandular tissue in the male breast. This condition is thought to be caused by the imbalance between estrogen action relative to androgen action at the breast tissue level. Bilateral gynecomastia is frequently found in the neonatal period, early in puberty, and with increasing age. Prepubertal unilateral gynecomastia in the absence of endocrine abnormalities is extremely rare, with only a few cases in literature. PRESENTATION OF CASE: We present an otherwise healthy boy of 12 years old with unilateral breast masses. No abnormalities were found on ultrasonography and on all endocrine parameters. Treatment consisted in a new "modified" Webster technique. DISCUSSION: The results confirmed validity of this technique in terms of esthetic and functional results, and patient satisfaction. Atypical presentations of gynecomastia are often not recognized. The main pathophysiology of gynecomastia is alteration in the balance between the stimulatory effect of estrogen and the inhibitory effects of androgens on the development of the breast. If there is no causal treatment, surgical resection is the therapy of first choice. CONCLUSION: The exact mechanism of unilateral gynecomastia formation in our case is unclear. The evaluation of unilateral gynecomastia can therefore be complex. In conclusion, the surgical treatment of unilateral gynecomastia requires an individual approach, based on an appropriate diagnostic algorithm.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4275969/

Record 1250

pmcid: PMC4129307

pmid: 25121023

doi: 10.7860/JCDR/2014/7920.4507

title: Clinical examination allied to ultrasonography in the assessment of new onset gynaecomastia: an observational study.

authors

- Athwal RK
- Donovan R
- Mirza M

journal: Journal of clinical and diagnostic research : JCDR

journalAbbr: J Clin Diagn Res

year: 2014

pubdate: 2014 Jun

volume: 8

issue: 6

pages: NC09-11

abstract: AIM: New onset gynaecomastia is a relatively common presentation to breast surgical services. The main aim of clinical and radiological evaluation is to exclude the presence of concurrent breast cancer. There exists much variability in the clinical assessment of male patients presenting with new onset gynaecomastia. MATERIALS AND METHODS: In the presented pilot study, all the male patients presenting with new onset gynaecomastia to our department over a period of two years were studied. RESULTS: Fifty three patients presented with new onset gynaecomastia during the study period. Clinical examination allied with ultrasonography confirmed benign breast disease in 50 patients with only three patient requiring breast biopsy to exclude malignancy. We detail the efficacy of utilising clinical examination in conjunction with ultrasonography to evaluate new onset gynaecomastia. CONCLUSIONS: We show that clinical examination used in conjunction with ultrasonography is both highly sensitive and specific for detecting male breast cancer in patients presenting with new onset gynaecomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4129307/

Record 1251

pmcid: PMC3997862

pmid: 24808966

doi: 10.1155/2014/815896

title: Congestive Heart Failure versus Inflammatory Carcinoma in Breast.

authors

- Alikhassi A
- Omranipour R
- Alikhassy Z

journal: Case reports in radiology

journalAbbr: Case Rep Radiol

year: 2014

pubdate: 2014

volume: 2014

issue:

pages: 815896

abstract: Inflammatory breast cancer is a rare highly malignant form of breast cancer. Clinical signs and symptoms with histologic examination usually confirm the diagnosis. There are rare reports of breast edema of congestive heart failure which were difficult to differentiate from inflammatory carcinoma. The differential becomes more difficult when congestive heart failure is associated with unilateral breast edema. We present a case of a 70-year-old woman with congestive heart failure associated with unilateral breast edema and skin thickening simulating inflammatory breast carcinoma on mammography.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3997862/

Record 1252

pmcid: PMC4075620

pmid: 24814851

doi: 10.1212/WNL.0000000000000507

title: Correlation of clinical and molecular features in spinal bulbar muscular atrophy.

authors

- Fratta P
- Nirmalananthan N
- Masset L
- Skorupinska I
- Collins T
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- Fisher EM
- Greensmith L
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journal: Neurology

journalAbbr: Neurology

year: 2014

pubdate: 2014 Jun 10

volume: 82

issue: 23

pages: 2077-84

abstract: OBJECTIVES: To characterize the clinical and genetic features of spinal bulbar muscular atrophy (SBMA), a rare neurodegenerative disorder caused by the expansion of a CAG repeat in the first exon of the androgen receptor gene, in the United Kingdom. METHODS: We created a national register for SBMA in the United Kingdom and recruited 61 patients between 2005 and 2013. In our cross-sectional study, we assessed, by direct questioning, impairment of activities of daily living (ADL) milestones, functional rating, and subjective disease impact, and performed correlations with both CAG repeat size and degree of somatic mosaicism. Ten patients were deceased, 46 patients participated in the study, and 5 declined. RESULTS: Subjects had an average age at onset of 43.4 years, and weakness onset most frequently occurred in the lower limbs (87%). Impaired mobility was the most frequently reported problem by patients, followed by bulbar dysfunction. Age distribution of the impairment of ADL milestones showed remarkable overlap with a Japanese study. We have identified a significant correlation between the number of CAG repeats and both age at onset and ADL milestones. Somatic mosaicism also showed a correlation with CAG expansion size and age at onset. CONCLUSIONS: Clinical features in SBMA show a substantial overlap when comparing populations with different genetic backgrounds. This finding has major implications, because multicenter trials will be necessary to obtain sufficient power in future clinical trials. Clinical-genetic correlations are strong in SBMA and should inform any clinical research strategy in this condition.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4075620/

Record 1253

pmcid: PMC4093982

pmid: 24872223

doi: 10.3945/jn.113.188318

title: Dietary calcium requirements do not differ between Mexican-American boys and girls.

authors

- Palacios C
- Martin BR
- McCabe GP
- McCabe L
- Peacock M
- Weaver CM

journal: The Journal of nutrition

journalAbbr: J Nutr

year: 2014

pubdate: 2014 Aug

volume: 144

issue: 8

pages: 1167-73

abstract: Mexican Americans are an understudied ethnic group for determinants of bone health, although the risk of age-related osteoporosis is high in this rapidly growing sector of the U.S. population. Thus, the objective

of the present study was to establish the dietary calcium requirements for bone health in Mexican-American adolescents by measuring calcium retention calculated from balance in response to a range of dietary calcium intakes and to determine predictors of skeletal calcium retention. Adolescents aged 12-15 y were studied twice on paired calcium intakes ranging from 600 to 2300 mg/d using randomized-order, crossover 3-wk balance studies. Skeletal calcium retention was calculated as dietary calcium intake minus calcium excreted in feces and urine over the last 2 wk of balance. A linear model was developed to explain the variation in calcium retention. Boys (n = 20) were taller and had higher lean mass, usual dietary calcium intake, bone mineral content, and serum alkaline phosphatase compared with girls, whereas girls (n = 20) had higher Tanner scores and greater fat mass. Calcium retention increased with calcium intake ($P < 0.0001$) and did not differ by sex ($P = 0.66$). In boys and girls considered together, calcium intake explained 33% of the variation in calcium retention. Serum alkaline phosphatase explained an additional 11% of the variation in calcium retention. Other variables measured, including the urine N-telopeptide of type I collagen/creatinine ratio, Tanner score, serum parathyroid hormone and 25-hydroxyvitamin D, weight, height, and body mass index, did not contribute to the variance in calcium retention. In adolescence, calcium retention in both Mexican-American boys and girls was higher than determined previously in adolescent nonHispanic white girls. This trial was registered at clinicaltrials.gov as NCT01277185.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4093982/

Record 1254

pmcid: PMC4229279

pmid: 25426403

doi: 10.1097/GOX.0000000000000183

title: Diffusion and perfusion: the keys to fat grafting.

authors

- Khouri RK Jr
- Khouri RE
- Lujan-Hernandez JR
- Khouri KR
- Lancerotto L
- Orgill DP

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2014

pubdate: 2014 Sep

volume: 2

issue: 9

pages: e220

abstract: BACKGROUND: Fat grafting is now widely used in plastic surgery. Long-term graft retention can be unpredictable. Fat grafts must obtain oxygen via diffusion until neovascularization occurs, so oxygen delivery may be the overarching variable in graft retention. METHODS: We studied the peer-reviewed literature to determine which aspects of a fat graft and the microenvironment surrounding a fat graft affect oxygen delivery and created 3 models relating distinct variables to oxygen delivery and graft retention. RESULTS: Our models confirm that thin microribbons of fat maximize oxygen transport when injected into a large, compliant, well-vascularized recipient site. The "Microribbon Model" predicts that, in a typical human, fat injections larger than 0.16 cm in radius will have a region of central necrosis. Our "Fluid Accommodation Model" predicts that once grafted tissues approach a critical interstitial fluid pressure of 9 mm Hg, any additional fluid will drastically increase interstitial fluid pressure and reduce capillary perfusion and oxygen delivery. Our "External Volume Expansion Effect Model" predicts the effect of vascular changes induced by preoperative external volume

expansion that allow for greater volumes of fat to be successfully grafted. CONCLUSIONS: These models confirm that initial fat grafting survival is limited by oxygen diffusion. Preoperative expansion increases oxygen diffusion capacity allowing for additional graft retention. These models provide a scientific framework for testing the current fat grafting theories.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4229279/

Record 1255

pmcid: PMC4276584

pmid: 25562023

doi:

title: Effect of pubertal suppression on linear growth and body mass index; a two-year follow-up in girls with genetic short stature and rapidly progressive puberty.

authors

- Karamizadeh Z
- Amirhakimi A
- Amirhakimi G

journal: Iranian journal of pediatrics

journalAbbr: Iran J Pediatr

year: 2014

pubdate: 2014 Jun

volume: 24

issue: 3

pages: 293-9

abstract: OBJECTIVE: Gonadotropin-Releasing Hormone agonists (GnRHa) are used to improve the final adult height in short stature children. There are limited studies which address the potential side effect of these agents: excessive weight gain. We have followed girls with rapidly progressive puberty receiving GnRHa and results were focused on the effect of treatment on final height, weight and body mass index Methods: Thirty girls between 8.5 and 12 years with short stature and predicted adult height of less than 155 cm were enrolled in the study. All had rapidly progressive puberty. Weight and height measurements were done at the beginning of treatment, 6 and 12 months after starting and 6 and 12 months after the cessation of treatment. Bone age and stages of puberty were estimated at the beginning of treatment, after 12 months of starting and 12 months after the treatment was stopped. Findings : Predicted adult height (PAH) changes during treatment were not significant. There was no significant difference between final height and weight according to the body mass index (BMI), PAH or bone age. CONCLUSION: We conclude that girls with genetic short stature and rapidly progressive puberty will not benefit receiving a one-year course of GnRHa and there is no significant difference between the final height and final weigh among children according to BMI.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4276584/

Record 1256

pmcid: PMC3935839

pmid: 24586518

doi: 10.1371/journal.pone.0089088

title: Effects of switching from stavudine to raltegravir on subcutaneous adipose tissue in HIV-infected patients with HIV/HAART-associated lipodystrophy syndrome (HALS). A clinical and molecular study.

authors

- Domingo P
- Gutierrez Mdel M
- Gallego-Escuredo JM
- Torres F
- Mateo GM
- Villarroya J
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journal: PloS one

journalAbbr: PLoS One

year: 2014

pubdate: 2014

volume: 9

issue: 2

pages: e89088

abstract: HIV-1/HAART-associated lipodystrophy syndrome (HALS) has been associated with exposure to stavudine (d4T) through mitochondrial dysfunction. We performed a 48-week study to assess the effects of switching from d4T to raltegravir (RAL) on metabolic and fat molecular parameters of patients with HALS. Forty-two patients with HALS and a median exposure to d4T > 7 years were switched to RAL and followed for 48 weeks. Fasting metabolic tests, HIV RNA, CD4 cell count, and fat measured by DEXA were obtained at baseline and week 48. mtDNA and gene transcripts for PPAR gamma, adiponectin, cytochrome b, Cox IV, TNF alpha, MCP-1 and CD68 were assessed in paired subcutaneous fat tissue biopsies. Lipid parameters, fasting glucose, insulin, and HOMA-IR did not change significantly. Whole body fat (P = 0.0027) and limb fat mass (P<0.0001) increased from baseline. Trunk/limb fat ratio (P = 0.0022), fat mass ratio (P = 0.0020), fat mass index (P = 0.0011) and percent leg fat normalized to BMI (P<0.0001) improved after 48 weeks. Relative abundance of mtDNA, expression of PPAR gamma, adiponectin, Cyt b, and MCP-1 genes increased, whereas Cox IV, TNF alpha, and CD68 did not change significantly from baseline. Switching from d4T to RAL in patients with HALS is associated with an increase in limb fat mass and an improvement in markers of adipocyte differentiation and mitochondrial function in SAT.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3935839/

Record 1257

pmcid: PMC4266866

pmid: 25538875

doi: 10.4103/2230-8210.145055

title: Endocrine check-up in adolescents and indications for referral: A guide for health care providers.

authors

- De Sanctis V
- Soliman AT
- Fiscina B
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- Elalaily R
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journal: Indian journal of endocrinology and metabolism

journalAbbr: Indian J Endocrinol Metab

year: 2014

pubdate: 2014 Nov

volume: 18

issue: Suppl 1

pages: S26-38

abstract: The American Academy of Pediatrics recommends that young people between the ages of 11 and 21 years should be seen annually by their pediatricians, since annual checkups can be an important opportunity for health evaluation and anticipatory guidance. Parents of infants and young children are accustomed to regularly visiting a pediatrician for their child's checkups. Unfortunately, when children reach the teen years, these annual checkups may decrease in frequency. In routine check-ups and medical office visits, particular attention should be paid to the possibility of a developmental or endocrine disorder. Early diagnosis and treatment may prevent medical complications in adulthood and foster age-appropriate development. Our purpose is to acquaint readers with the concept, based on current scientific understanding, that some endocrine disorders may be associated with a wide range of deleterious health consequences including an increased risk of hypertension and hyperlipidemia, increased risk of coronary artery disease, type 2 diabetes, significant anxiety and lack of self-esteem. Understanding the milestones and developmental stages of adolescence is essential for pediatricians and all other health providers who care for adolescents. Treating adolescents involves knowledge of a variety of medical, social and legal information; in addition, close working relationships must be established within the adolescent's network to create an effective care system. In summary, we underline the importance of a periodic endocrine checkup in adolescents in order to identify endocrine problems early and develop an approach to treatment for those patients who need help during this time. Indications for endocrine referral for professional and other healthcare providers are also included. These lists are clearly not intended to be comprehensive, but will hopefully serve as a guide for specific clinical circumstances.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4266866/

Record 1258

pmcid: PMC4127841

pmid: 25120299

doi: 10.4103/0971-4065.132021

title: Extending the longevity of a complicated arteriovenous fistula using endovascular intervention.

authors

- Pawar B
- Fernandes K
- Sajiv CT

journal: Indian journal of nephrology

journalAbbr: Indian J Nephrol

year: 2014

pubdate: 2014 May

volume: 24

issue: 3

pages: 185-8

abstract: A brachiocephalic arteriovenous fistula was complicated by a central venous stenosis, which could not be relieved. A cephalojugular bypass was performed using an interpositoned graft, which later developed tight stenoses at both ends of the graft. This was successfully treated with endovascular intervention, extending the longevity of the vascular access.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4127841/

Record 1259

pmcid: PMC4060393

pmid: 24987424

doi: 10.1155/2014/102579

title: Factors affecting parent-adolescent discussion on reproductive health issues in Harar, eastern Ethiopia: a cross-sectional study.

authors

- Yadeta TA
- Bedane HK
- Tura AK

journal: Journal of environmental and public health

journalAbbr: J Environ Public Health

year: 2014

pubdate: 2014

volume: 2014

issue:

pages: 102579

abstract: BACKGROUND: Open family discussion on reproductive health (RH) issues often leads to increased awareness on RH matters and reduces risky behaviors among adolescents. This study was conducted to assess factors affecting parent-adolescent discussion on RH issues in Harar, Ethiopia. METHODS: A cross-sectional survey using face to face interview supplemented with focus group discussion (FGD) was conducted on 751 randomly selected parents of 10-19-year-old adolescents. Data was analyzed using SPSS version 15. RESULTS: More than one-fourth (28.76%) of parents reported discussing RH issues with their adolescents during the last six months. In the logistic regression, parents who have demonstrated good RH knowledge and positive attitude towards RH were almost six times and seventy percent (AOR 5.69, 95% CI: 3.67-8.82; AOR 1.70, 95% CI: 1.08-2.68) higher in discussing RH with their adolescents than their counterparts, respectively. CONCLUSION: Parent-adolescent discussion about RH issues rarely occurs and is bounded by lack of knowledge, sociocultural norms, and parental concern that discussion would encourage premarital sex. Reproductive health programs should target on improving awareness of parents and addressing sociocultural norms surrounding reproductive health issues.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4060393/

Record 1260

pmcid: PMC3998851

pmid: 24790468

doi: 10.2147/CMAR.S60006

title: Genetic factors and breast cancer laterality.

authors

- Amer MH

journal: Cancer management and research

journalAbbr: Cancer Manag Res

year: 2014

pubdate: 2014

volume: 6

issue:

pages: 191-203

abstract: BACKGROUND: Women are more likely to develop cancer in the left breast than the right. Such laterality may influence subsequent management, especially in elderly patients with heart disease who may require radiation therapy. The purpose of this study was to explore possible factors for such cancer laterality. METHODS: In this work, clinical data for consecutive patients with histologically confirmed breast cancer were reviewed, with emphasis on clinical presentation and family history. RESULTS: Between 2005 and 2012, 687 patients with breast cancer were seen. Two women with incomplete data and eleven men were excluded. In total, 343 (50.9%) patients presented with left breast cancer, 311 (46.1%) with right breast cancer, and 20 (3.0%) with simultaneous bilateral malignancy. There were no significant differences between the three groups, especially in regards to clinical presentation and tumor characteristics. A total of 622 (92.3%) patients had unilateral primary, 20 (3.0%) had simultaneous bilateral, and 32 (4.7%) had metachronous primary breast cancer with subsequent contralateral breast cancer after 7.5-236 months. The worst 10-year survival was for bilateral simultaneous (18%) compared with unilateral (28%) and metachronous primaries (90%). There were no differences in survival in relation to breast cancer laterality, handedness, and presence or absence of a family history of cancer. There were significant similarities between patients and first-degree relatives in regards to breast cancer laterality, namely same breast (30/66, 45.5%), opposite breast (9/66, 13.6%), and bilateral cancer (27/66, 40.9, P=0.01163). This was more evident among patients and their sisters (17/32, 53.1%) or mothers (11/27, 40.7%, P=0.0689). There were also close similarities in relation to age at initial diagnosis of cancer for patients and their first-degree relatives for age differences of ≤5 years (48/166, 28.9%), 6-10 years (34/166, 20.5%), and >11 years (84/166, 50.6%, P=0.12065). CONCLUSION: High similarities between patients and their first-degree relatives in regards to cancer laterality and possibly age at initial diagnosis of cancer may suggest an underlying inherited genetic predisposition.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3998851/

Record 1261

pmcid: PMC4222024

pmid: 25389498

doi: 10.5812/ircmj.9531

title: Giant fibroadenoma growing rapidly during pregnancy.

authors

- Karagulle E
- Turk E
- Erinanc OH
- Moray G

journal: Iranian Red Crescent medical journal

journalAbbr: Iran Red Crescent Med J

year: 2014

pubdate: 2014 Aug

volume: 16

issue: 8

pages: e9531

abstract: INTRODUCTION: Giant fibroadenoma is a rare disease with unknown etiology. During pregnancy, fibroadenomas increase in size and may show lactational histologic changes. High concentrations of estrogen, progesterone, and prolactin promote the ductal growth and formation of tubuloalveolar structures. This may be a reason for the significant enlargement in this period. CASE PRESENTATION: We presented a case of giant fibroadenoma, first detected at the onset of pregnancy, which grew rapidly and was excised surgically two months after the birth. There was no marked deformity in the breast nor a need to reconstruct it, despite the giant mass was excised and the mother was lactating. DISCUSSION: We presented a rare case of giant fibroadenoma in a lactating woman. A progressively growing mass in breast can lead to structural damages. The current management approach for giant fibroadenomas is still surgical excision.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4222024/

Record 1262

pmcid: PMC4204242

pmid: 25336799

doi: 10.4103/0971-9261.141995

title: Gonadal germ cell tumors in children and adolescents.

authors

- Cecchetto G

journal: Journal of Indian Association of Pediatric Surgeons

journalAbbr: J Indian Assoc Pediatr Surg

year: 2014

pubdate: 2014 Oct

volume: 19

issue: 4

pages: 189-94

abstract: UNLABELLED: PEDIATRIC GERM CELL TUMORS (GCT) ARE RARE TUMORS: 80% are benign, 20% malignant (2-3% of all malignant pediatric tumors). The gonadal sites (ovary and testis) account for 40% of cases. OVARIAN GCTS: Represent 30% of GCTs and 70% of neoplastic ovarian masses, being the most common ovarian neoplasms in children and teenagers. Benign and immature forms (teratomas) constitute about 80% of all ovarian GCTs, malignant forms represent 20% increasing during adolescence. The most common malignant entity in children is the yolk sac tumors (YST); dysgerminoma is frequent during adolescence and being bilateral in 10% of cases. Presentation is similar in malignant and benign lesions; abdominal pain (70-80%) and lower abdominal mass are common symptoms. Evaluation of alpha-fetoprotein (αFP) or beta subunit of human chorionic gonadotropin (βHCG) is essential to address the nature of the tumors: Their elevation means presence of malignancy. Surgery includes intraoperative staging procedures and requires ovariectomy or ovarosalpingectomy for malignant lesions, but may be conservative in selected benign tumors. Since malignant GCTs are very chemosensitive, primary chemotherapy is recommended in metastatic or locally advanced tumors. TESTICULAR GCT: Represent 10% of pediatric GCT, and about 30% of malignant GCT with two age peaks: Children <3 years may experience mature teratoma and malignant GCTs, represented almost exclusively by YST, while adolescents may also show seminomas or other mixed tumors. The main clinical feature is a painless scrotal mass. Surgery represents the cornerstone of the management of testicular GCTs, with an inguinal approach and a primary high orchidectomy for malignant tumors, while a testis-sparing surgery

can be considered for benign lesions. A retroperitoneal lymph node (LN) biopsy may be necessary to define the staging when the involvement of retroperitoneal LN is uncertain at imaging investigations. CONCLUSION: Patients with gonadal malignant GCTs fare better than those with extragonadal mediastinal germ cell tumors (MGCTs) and survival rate exceeds 90% in localized forms. Chemotherapy has significantly improved the outcome of malignant forms since the introduction of platinum based regimens. The surgical procedure has to be performed in agreement with the ongoing protocols.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4204242/

Record 1263

pmcid: PMC3987263

pmid: 24741509

doi: 10.4103/2230-8210.129104

title: Gynecomastia: Clinical evaluation and management.

authors

- Cuhaci N
- Polat SB
- Evranos B
- Ersoy R
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journal: Indian journal of endocrinology and metabolism

journalAbbr: Indian J Endocrinol Metab

year: 2014

pubdate: 2014 Mar

volume: 18

issue: 2

pages: 150-8

abstract: Gynecomastia is the benign enlargement of male breast glandular tissue and is the most common breast condition in males. At least 30% of males will be affected during their life. Since it causes anxiety, psychosocial discomfort and fear of breast cancer, early diagnostic evaluation is important and patients usually seek medical attention. Gynecomastia was reported to cause an imbalance between estrogen and androgen action or an increased estrogen to androgen ratio, due to increased estrogen production, decreased androgen production or both. Evaluation of gynecomastia must include a detailed medical history, clinical examination, specific blood tests, imaging and tissue sampling. Individual treatment requirements can range from simple reassurance to medical treatment or even surgery. The main aim of any intervention is to relieve the symptoms and exclude other etiological factors.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3987263/

Record 1264

pmcid: PMC4108257

pmid: 25083134

doi: 10.2147/TCRM.S35075

title: HIV-associated lipodystrophy: a review from a Brazilian perspective.

authors

- Alves MD

- Brites C
- Sprinz E

journal: Therapeutics and clinical risk management

journalAbbr: Ther Clin Risk Manag

year: 2014

pubdate: 2014

volume: 10

issue:

pages: 559-66

abstract: The prognosis of human immunodeficiency virus (HIV)-infected individuals has dramatically improved worldwide since the introduction of highly antiretroviral therapy. Nevertheless, along with the decrease in mortality, several body modifications not initially related to HIV infection have been reported. Disorders in lipid and glucose metabolism, accompanied by body shape abnormalities and alterations in fat distribution, began to be described. A syndrome, named "HIV-associated lipodystrophy syndrome", was coined to classify these clinical spectrum aspects. This syndrome involves not only metabolic alterations but also fat redistribution, with lipoatrophy due to subcutaneous fat loss (predominantly in the face and lower limbs) and lipohypertrophy related to central fat gain. These changes in body shape are very important to be recognized, as they are associated with worse morbidity and mortality. Self-esteem difficulties related to body alterations might lead to treatment failures due to medication adherence problems. Moreover, these alterations have been associated with an increased risk of cardiovascular events. Therefore, it is extremely important to identify this syndrome early in order to provide an even better quality of life for this population, as the clinical approach is not easy. Treatment change, medications to treat dyslipidemia, and surgical intervention are instruments to be used to try to correct these abnormalities. The aim of this study is to review clinical presentation, diagnosis, and management of body shape and metabolic complications of HIV infection from a Brazilian perspective, a medium income country with a large number of patients on antiretroviral therapy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4108257/

Record 1265

pmcid: PMC4474130

pmid: 25350171

doi: 10.1308/003588414X13946184903162

title: Iatrogenic arteriovenous fistula of the breast as a complication of core needle biopsy.

authors

- Haider MH
- Satpathy A
- Abou-Samra W

journal: Annals of the Royal College of Surgeons of England

journalAbbr: Ann R Coll Surg Engl

year: 2014

pubdate: 2014 Nov

volume: 96

issue: 8

pages: e20-2

abstract: Ultrasonography guided core biopsy has become the standard of care in the assessment of breast cancer. Although the procedure is associated with low rates of complications, we present a case report of a rare

complication of the procedure that has been reported only once previously in the medical literature.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4474130/

Record 1266

pmcid: PMC3936263

pmid: 24513565

doi: 10.2196/jmir.2694

title: In the words of the medical tourist: an analysis of Internet narratives by health travelers to Turkey.

authors

- Ozan-Rafferty ME
- Johnson JA
- Shah GH
- Kursun A

journal: Journal of medical Internet research

journalAbbr: J Med Internet Res

year: 2014

pubdate: 2014 Feb 6

volume: 16

issue: 2

pages: e43

abstract: BACKGROUND: Patients regularly travel to the West for advanced medical care, but now the trend is also shifting in the opposite direction. Many people from Western countries now seek care outside of their country. This phenomenon has been labeled medical tourism or health travel. Information regarding health travelers' actual outcomes, experiences, and perceptions is lacking or insufficient. However, advanced Internet technology and apps provide information on medical tourism and are a vehicle for patients to share their experiences. Turkey has a large number of internationally accredited hospitals, is a top tourism destination, and is positioning itself to attract international patients. OBJECTIVE: The objective of this research was to identify the important individual characteristics of health travelers, outline the push and pull factors for seeking health care in Turkey, identify satisfaction with the outcomes and the results of these individuals' treatments, and note positive and negative factors influencing their perceptions and overall experiences about patients' health travel. METHODS: This research uses qualitative data from Internet narratives of medical tourists to Turkey. Ethical considerations of using Internet narratives were reviewed. Narratives for analysis were obtained by using the Google search engine and using multiple search terms to obtain publicly posted blogs and discussion board postings of health travelers via purposeful sampling. Narratives were included if they were written in English, described travel to Turkey for health care, and were publicly accessible. Exclusion criteria included narratives that were on medical tourism facilitator or provider promotional websites, not in English, and did not describe an experience of a medical tourist. Medical tourists' written words were analyzed in an iterative analytic process using narrative analysis theory principles. Three stages of coding (open, axial, and selective) were conducted to identify characteristics and themes using qualitative analysis software. RESULTS: The narrative posts of 36 individuals undergoing 47 procedures who traveled to Turkey for medical care between 2007 and 2012 were analyzed. The narratives came from 13 countries, not including the narratives for which patient origin could not be determined. Travelers were predominantly from Europe (16/36, 44%) and North America (10/36, 28%). Factors driving travelers away from their home country (push factors) were cost and lack of treatment options or insufficient insurance coverage in their home country. Leading factors attracting patients to destination (pull factors) were lower costs, physician's expertise and responsiveness, and familiarity or interest in Turkey. Health travelers to Turkey were generally satisfied with the outcomes of their procedures and care provided by their physicians, many noting intent to return. Communication challenges, food, transportation, and gaps in customer service emerged as key areas for improvement. CONCLUSIONS: This analysis provides an understanding of

the insights of medical tourists through the words of actual health travelers. This nonintrusive methodology provides candid insights of common themes of health travelers and may be applied to study other patient experiences. The findings of this research expands the body of knowledge in medical tourism and serves as a platform for further qualitative and quantitative research on health travelers' experiences.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3936263/

Record 1267

pmcid: PMC4216802

pmid: 25368678

doi: 10.7150/jca.9835

title: In vivo lymphatic imaging of a human inflammatory breast cancer model.

authors

- Agollah GD
- Wu G
- Sevick-Muraca EM
- Kwon S

journal: Journal of Cancer

journalAbbr: J Cancer

year: 2014

pubdate: 2014

volume: 5

issue: 9

pages: 774-83

abstract: BACKGROUND: Inflammatory breast cancer (IBC) remains the most aggressive type of breast cancer with the greatest potential for metastasis and as a result, the highest mortality rate. IBC cells invade and metastasize through dermal lymphatic vessels; however, it is unknown how lymphatic drainage patterns change during IBC growth and metastasis. Herein, we non-invasively and longitudinally imaged lymphatics in an animal model of IBC using near-infrared fluorescence (NIRF) imaging. MATERIALS AND METHODS: Mice were imaged in vivo prior to, and up to 11 weeks after subcutaneous or orthotopic inoculation of human IBC SUM149 cells, which were stably transfected with infrared fluorescence protein (iRFP) gene reporter (SUM149-iRFP), following intradermal (i.d.) injection of indocyanine green (ICG). RESULTS: Fluorescence images showed well-defined lymphatic vessels prior to SUM149-iRFP inoculation. However, altered lymphatic drainage patterns including rerouting of lymphatic drainage were detected in mice with SUM149-iRFP, due to lymphatic obstruction of normal lymphatic drainages caused by tumor growth. In addition, we observed tortuous lymphatic vessels and extravasation of ICG-laden lymph in mice with SUM149-iRFP. We also observed increased and dilated fluorescent lymphatic vessels in the tumor periphery, which was confirmed by ex vivo immunohistochemical staining of lymphatic vessels. CONCLUSIONS: Our pre-clinical studies demonstrate that non-invasive NIRF imaging can provide a method to assess changes in lymphatic drainage patterns during IBC growth and metastasis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4216802/

Record 1268

pmcid: PMC4120190

pmid: 24529621

doi: 10.1016/j.jpeds.2013.12.035

title: In-hospital formula use increases early breastfeeding cessation among first-time mothers intending to

exclusively breastfeed.

authors

- Chantry CJ
- Dewey KG
- Peerson JM
- Wagner EA
- Nommsen-Rivers LA

journal: The Journal of pediatrics

journalAbbr: J Pediatr

year: 2014

pubdate: 2014 Jun

volume: 164

issue: 6

pages: 1339-45.e5

abstract: OBJECTIVE: To evaluate in-hospital formula supplementation among first-time mothers who intended to exclusively breastfeed and determined if in-hospital formula supplementation shortens breastfeeding duration after adjusting for breastfeeding intention. STUDY DESIGN: We assessed strength of breastfeeding intentions prenatally in a diverse cohort of expectant primiparae and followed infant feeding practices through day 60. Among mothers planning to exclusively breastfeed their healthy term infants for ≥1 week, we determined predictors, reasons, and characteristics of in-hospital formula supplementation, and calculated the intention-adjusted relative risk (ARR) of not fully breastfeeding days 30-60 and breastfeeding cessation by day 60 with in-hospital formula supplementation (n = 393). RESULTS: Two hundred ten (53%) infants were exclusively breastfed during the maternity stay and 183 (47%) received in-hospital formula supplementation. The most prevalent reasons mothers cited for in-hospital formula supplementation were: perceived insufficient milk supply (18%), signs of inadequate intake (16%), and poor latch or breastfeeding (14%). Prevalence of not fully breastfeeding days 30-60 was 67.8% vs. 36.7%, ARR 1.8 (95% CI, 1.4-2.3), in-hospital formula supplementation vs exclusively breastfed groups, respectively, and breastfeeding cessation by day 60 was 32.8% vs. 10.5%, ARR 2.7 (95% CI, 1.7-4.5). Odds of both adverse outcomes increased with more in-hospital formula supplementation feeds (not fully breastfeeding days 30-60, P = .003 and breastfeeding cessation, P = .011). CONCLUSIONS: Among women intending to exclusively breastfeed, in-hospital formula supplementation was associated with a nearly 2-fold greater risk of not fully breastfeeding days 30-60 and a nearly 3-fold risk of breastfeeding cessation by day 60, even after adjusting for strength of breastfeeding intentions. Strategies should be sought to avoid unnecessary in-hospital formula supplementation and to support breastfeeding when in-hospital formula supplementation is unavoidable.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4120190/

Record 1269

pmcid: PMC4116310

pmid: 25114624

doi:

title: Is routine pathological evaluation of tissue from gynecomastia necessary? A 15-year retrospective pathological and literature review.

authors

- Senger JL
- Chandran G
- Kanthan R

journal: Plastic surgery (Oakville, Ont.)

journalAbbr: Plast Surg (Oakv)

year: 2014

pubdate: 2014 Summer

volume: 22

issue: 2

pages: 112-6

abstract: OBJECTIVE: To reconsider the routine plastic surgical practice of requesting histopathological evaluation of tissue from gynecomastia. METHOD: The present study was a retrospective histopathological review (15-year period [1996 to 2012]) involving gynecomastia tissue samples received at the pathology laboratory in the Saskatoon Health Region (Saskatchewan). The Laboratory Information System (LIS) identified all specimens using the key search words "gynecomastia", "gynaecomastia", "gynecomazia" and "gynaecomazia". A literature review to identify all cases of incidentally discovered malignancies in gynecomastia tissue specimens over a 15-year period (1996 to present) was undertaken. RESULTS: The 15-year LIS search detected a total of 452 patients that included two cases of pseudogynecomastia (0.4%). Patients' age ranged from five to 92 years and 43% of the cases were bilateral (28% left sided, 29% right sided). The weight of the specimens received ranged from 0.2 g to 1147.2 g. All cases showed no significant histopathological concerns. The number of tissue blocks sampled ranged from one to 42, averaging four blocks/case (approximately $105/case), resulting in a cost of approximately $3,200/year, with a 15-year expenditure of approximately $48,000. The literature review identified a total of 15 incidental findings: ductal carcinoma in situ (12 cases), atypical ductal hyperplasia (two cases) and infiltrating ductal carcinoma (one case). CONCLUSIONS: In the context of evidence-based literature, and because no significant pathological findings were detected in this particular cohort of 452 cases with 2178 slides, the authors believe it is time to re-evaluate whether routine histopathological examination of tissue from gynecomastia remains necessary. The current climate of health care budget fiscal restraints warrants reassessment of the current policies and practices of sending tissue samples of gynecomastia incurring negative productivity costs on routine histopathological examination.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4116310/

Record 1270

pmcid: PMC4128428

pmid: 25152642

doi:

title: Late unilateral hematoma after breast augmentation.

authors

- Peters W
- Fornasier V
- Howarth D

journal: Plastic surgery (Oakville, Ont.)

journalAbbr: Plast Surg (Oakv)

year: 2014

pubdate: 2014 Spring

volume: 22

issue: 1

pages: 18-21

abstract: BACKGROUND: Late hematoma after breast augmentation is a rare phenomenon with unproven

etiology. METHODS: Five patients presented with a late unilateral hematoma after receiving bilateral, submuscular, smooth, round, silicone gel implants for cosmetic breast augmentation. All patients underwent explantation and capsulectomy. Extensive clinical and histological analysis of all capsules was performed. RESULTS: All patients presented with progressive unilateral breast enlargement with at least a doubling of their breast size. Breast enlargement developed nine, 12, 14, 22 and 38 years, respectively, after initial implant insertion and in the absence of any known trauma. Patients presented for treatment one, three, four, nine and 12 months, respectively, after initially noticing breast enlargement. This enabled a sequential analysis of the events occurring on the surface of the capsule and within the structure of the capsule. Macroscopic and microscopic analysis of all capsules demonstrated multiple areas of recent and older hemorrhage, both within the structure of capsules and on the surface of the capsule. Continuing recurrent bouts of acute hemorrhage were observed even nine and 12 months after initial hematoma development. The position of medium-size vessels within the capsules corresponded to the sites of recurrent bleeding within capsules. CONCLUSIONS: Late hematomas presented from nine to 38 years after submuscular breast augmentation with a doubling of breast size. Late hematomas appeared to result from multiple recurrent bouts of bleeding within the structure of the capsules. This leads to recurrent bleeding on the surface of the capsules. Breast enlargement develops progressively over time rather than acutely. Continuing acute hemorrhage persisted up to one year after initial hematoma development. This progression suggests a process analogous to a chronic expanding hematoma that has been described in other areas of the body. Failure to remove a capsule could result in long-term continuing bleeding from vessels within the capsule.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4128428/

Record 1271

pmcid: PMC4137697

pmid: 25215252

doi: 10.1155/2014/610363

title: Management and follow-up of a case of gestational gigantomastia in a brazilian hospital.

authors

- Eler Dos Reis P
- Blunck Santos NQ
- Barbosa Pagio FA
- Chambô F
- Chambô D
- Chambô Filho A

journal: Case reports in obstetrics and gynecology

journalAbbr: Case Rep Obstet Gynecol

year: 2014

pubdate: 2014

volume: 2014

issue:

pages: 610363

abstract: Gigantomastia is a breast disorder that is associated with an exaggerated, rapid growth of the breasts, generally bilaterally. Since the pathology is rare and has seldom been described, its etiology has yet to be fully established, although there are speculations that a hormonal component may play an important role. Treatment is aimed at improving the clinical and psychological symptoms; however, the best therapeutic option varies from case to case. The present report describes a case of gestational gigantomastia seen at the Department of Obstetrics and Gynecology of the Hospital da Santa Casa de Misericórdia, Vitória, Espírito Santo, Brazil, in a primigravida in the second trimester of pregnancy. The report follows this patient from her diagnosis until the completion of treatment with a third and final surgical procedure.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4137697/

Record 1272

pmcid: PMC4138538

pmid: 25027062

doi: 10.1007/s11904-014-0219-7

title: Metabolic syndrome, diabetes, and cardiovascular risk in HIV.

authors

- Nix LM
- Tien PC

journal: Current HIV/AIDS reports

journalAbbr: Curr HIV/AIDS Rep

year: 2014

pubdate: 2014 Sep

volume: 11

issue: 3

pages: 271-8

abstract: HIV infection and its treatment have been associated with adipose tissue changes and disorders of glucose and lipid metabolism. The proportion of HIV-infected adults over the age of 50 is also growing placing HIV-infected adults at particular risk for metabolic perturbations and cardiovascular disease. The metabolic syndrome in HIV-infected adults has been increasingly studied but whether HIV is associated with greater risk remains unclear, likely because of the interplay of host, viral and antiretroviral factors that are associated with the components of the metabolic syndrome. The relationship between HIV and diabetes mellitus (DM) risk has also been debated. While the Framingham Risk Score is a well-accepted measure of 10-year cardiovascular risk in the general population, it may not accurately predict risk in the HIV setting due to HIV-related factors such as inflammation that are not accounted for. We summarize the recent literature on metabolic syndrome, DM, and cardiovascular risk in HIV-infected adults.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4138538/

Record 1273

pmcid: PMC4086161

pmid: 24742452

doi: 10.1016/j.neuron.2014.04.005

title: Muscle matters in Kennedy's disease.

authors

- Rinaldi C
- Bott LC
- Fischbeck KH

journal: Neuron

journalAbbr: Neuron

year: 2014

pubdate: 2014 Apr 16

volume: 82

issue: 2

pages: 251-3

abstract: Polyglutamine expansion in the androgen receptor causes Kennedy's disease. Two recent reports, Cortes et al. (2014) in this issue of Neuron and Lieberman et al. (2014) in Cell Reports, raise the possibility that targeting expression of the mutant protein in skeletal muscle, instead of the nervous system, may mitigate manifestations of this disorder.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4086161/

Record 1274

pmcid: PMC4420425

pmid: 26023473

doi:

title: Neonatal mastitis: a clinico-microbiological study.

authors

- Masoodi T
- Mufti GN
- Bhat JI
- Lone R
- Arshi S
- Ahmad SK

journal: Journal of neonatal surgery

journalAbbr: J Neonatal Surg

year: 2014

pubdate: 2014 Jan-Mar

volume: 3

issue: 1

pages: 2

abstract: OBJECTIVE: Neonatal breast hypertrophy is a common phenomenon in term infants, superadded infection can lead to mastitis and that can progress to breast abscess with short and long term detrimental effects. Our effort is to study the prevalence, risk factors, the current microbial profile and sensitivity pattern in these infections in order to suggest an optimal treatment plan for these patients. DESIGN: Case series. SETTING: Hospital based study conducted in Kashmir on the native population. DURATION: 2011 to 2013. MATERIALS AND METHODS: 32 neonates with features of mastitis or abscess were included in the study. Demographic and clinical data, laboratory work-up were recorded for all these patients in a patient form. Gram stain of the purulent nipple discharge or pus obtained on drainage was done and the specimens were culture plated. Antibiotic sensitivity was determined by disk diffusion and categorized by current Clinical and Laboratory Standards Institute (CLSI) guidelines. RESULTS: Most babies were full term, the age range was 6-48 days. Peak incidence for mastitis was in the 2nd week and for abscess in the 4th week. The ratio of male: female was 1:2 in the entire group, there was greater preponderance of female involvement with increasing age. Massage for expression of secretions a common practice in the study population had been done in 15 patients, especially in male babies. The babies were generally well and associated skin pustulosis was common. Laboratory workup showed polymorphonuclear leucocytosis and CRP positivity. Gram staining showed gram positive cocci in 13 patients and gram negative rods in 1 patient. Culture revealed Staphylococcus aureus in 18, E.col in 2, klebsiella in 1 patient and was sterile in 2 patients. Most strains of Staphylococcus aureus were resistant to macrolides and penicillins. Fifteen were methicillin sensitive and 3 were resistant but were sensitive to amikacin, ofloxacin and vancomycin. Gram negative rods were sensitive to, aminoglycosides, chloramphenicol, quinolones, piperacillin-tazobactum and cefoperazone-sulbactum, but were resistant to cephalosporins including

third generation cephalosporins. Treatment with oral antibiotic was not successful. Patients responded well to open drainage via a stab incision away from the breast mound; 4 patients were managed by repeated needle aspirations. IV antibiotics were prescribed in all patients for 2-5 days, followed by oral continuation therapy of 7-14 days. CONCLUSION: From our study, we can conclude that parental counseling to avoid massage, and early treatment for pustulosis is important to prevent mastitis. Intravenous antibiotics should be used for this condition guided by gram stain or culture sensitivity once available. Empirically a drug with good anti-staph cover may be instituted till appropriate reports are available. Incision drainage gives uniformly good results, though; multiple sittings of needle drainage may obviate the need for incision drainage. Therapy can be shifted to oral drugs once clinical improvement is seen.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4420425/

Record 1275

pmcid: PMC10564070

pmid: 24893234

doi: 10.1212/01.CON.0000450966.68828.45

title: Neurologic complications of nondiabetic endocrine disorders.

authors

- Ishii M

journal: Continuum (Minneapolis, Minn.)

journalAbbr: Continuum (Minneap Minn)

year: 2014

pubdate: 2014 Jun

volume: 20

issue: 3 Neurology of Systemic Disease

pages: 560-79

abstract: PURPOSE OF REVIEW: This article provides an overview of the neurologic complications found in the various endocrine disorders affecting adult patients. Specifically, disorders in pituitary hormones (prolactin, growth hormone, vasopressin, and oxytocin), thyroid hormones, adrenal hormones (glucocorticoids), and sex hormones (estrogen and testosterone) will be covered, with an emphasis on identifying the signs and symptoms in addition to diagnosing and managing these disorders. RECENT FINDINGS: Hyperthyroidism in the young was found to increase the risk for ischemic stroke in a recent prospective case-cohort study. The cognitive effects of hormonal therapy in postmenopausal women remain controversial, but a recent study found no benefit or risk in cognitive function when treating younger (50 to 55 years of age) postmenopausal women with hormonal therapy. SUMMARY: Endocrine disorders can cause various neurologic complications, from insidious myopathy to acute encephalopathy. Diagnosing the endocrine disorder as the cause of the neurologic impairment is essential, as treating the underlying hormonal dysfunction will often rapidly reverse the neurologic symptoms. Ongoing research is needed to further clarify the role of hormonal dysfunction in neurologic disorders.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10564070/

Record 1276

pmcid: PMC3986738

pmid: 24637309

doi: 10.4274/Jcrpe.1166

title: Patient with mutation in the matrix metalloproteinase 2 (MMP2) gene - a case report and review of the literature.

authors

- Ekbote AV
- Danda S
- Zankl A
- Mandal K
- Maguire T
- Ungerer K

journal: Journal of clinical research in pediatric endocrinology

journalAbbr: J Clin Res Pediatr Endocrinol

year: 2014

pubdate: 2014

volume: 6

issue: 1

pages: 40-6

abstract: Torg and Winchester syndromes and patients reported by Al-AqeelSawairi as well as nodulosis-arthropathy-osteolysis (NAO) patients, patients with multicentric NAO share autosomal recessive inheritance. The common presenting symptomatology includes progressive osteolysis chiefly affecting the carpal, tarsal and interphalangeal joints. Here, we report a patient with Torg syndrome. Torg syndrome is caused by homozygous or compound heterozygous mutations in the matrix metalloproteinase 2 (MMP2) gene. MMP2 codes for a gelatinase that cleaves type IV collagen, a major component of basement membrane. The clinical presentation of our patient included moderate osteolysis of the small joints of the hands and knees, hirsutism, nodulosis sparing the palms and soles, corneal opacities and mild facial dysmorphism without gum hypertrophy. Genetic analysis showed that the patient was homozygous for a novel base variant c538 G>A (p.D180N) in the MMP2 gene. Both parents were carriers of the same mutated variant. Our patient had some previously unreported endocrine manifestations such as premature thelarche and elevated follicle-stimulating hormone levels.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3986738/

Record 1277

pmcid: PMC4335432

pmid: 25709134

doi: 10.4314/gmj.v48i4.5

title: Pharmacoepidemiology of antiretroviral drugs in a teaching hospital in Lagos, Nigeria.

authors

- Oreagba IA
- Usman SO
- Olayemi SO
- Oshikoya KA
- Opanuga O
- Adeyemo TA
- Lesi OA
- Dodoo AN
- Akanmu AS

journal: Ghana medical journal

journalAbbr: Ghana Med J

year: 2014

pubdate: 2014 Dec

volume: 48

issue: 4

pages: 194-203

abstract: OBJECTIVE: Prescribing, adherence, and adverse drug events to HAART in a large antiretroviral programme in Lagos was evaluated. DESIGN: A retrospective 5 year open cohort study. SETTING: The AIDS Prevention Initiative in Nigeria (APIN) clinic at LUTH is one of the United States Presidential Emergency Plan for AIDS Relief (PEP-FAR) funded centers for HIV relief program in Nigeria Participants The case files of 390 patients on HAART and attending the APIN clinic were reviewed sequel to random selection. MAIN OUTCOME MEASURES: Demographics of the patients and pattern of antiretroviral (ARV) combination drugs prescribed were extracted from their case files. The details of the adverse drug events (ADEs) were extracted from drug toxicity forms regularly filled for each patient. A Chi-square test with Yates correction was used to determine the association between adherence and therapeutic outcome. RESULTS: A total of 2944 prescriptions were assessed. Zidovudine + lamivudine + nevirapine (35.87%) and stavudine + lamivudine + nevirapine (35.63%) were the most frequently prescribed combinations. Over 2000 ADEs were reported with cough (13.3%), fever (8.75%) and skin rashes (8.01%) being the most frequently reported. Drug adherence was associated with good therapeutic outcome ($\chi(2) = 115.60$, $p<0.0001$). CONCLUSIONS: Zidovudine + lamivudine + nevirapine was the most frequently prescribed ARV combination. Cough was the most frequently reported ADE. Interventions aimed at rational prescribing of ARV drugs and improving adherence to antiretroviral drugs is essential for good therapeutic outcome in the treatment of HIV infection.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4335432/

Record 1278

pmcid: PMC4256163

pmid: 25474635

doi: 10.1371/journal.pone.0113632

title: Physiologically-based toxicokinetic modeling of zearalenone and its metabolites: application to the Jersey girl study.

authors

- Mukherjee D
- Royce SG
- Alexander JA
- Buckley B
- Isukapalli SS
- Bandera EV
- Zarbl H
- Georgopoulos PG

journal: PloS one

journalAbbr: PLoS One

year: 2014

pubdate: 2014

volume: 9

issue: 12

pages: e113632

abstract: Zearalenone (ZEA), a fungal mycotoxin, and its metabolite zeranol (ZAL) are known estrogen agonists in mammals, and are found as contaminants in food. Zeranol, which is more potent than ZEA and comparable in potency to estradiol, is also added as a growth additive in beef in the US and Canada. This article presents the development and application of a Physiologically-Based Toxicokinetic (PBTK) model for ZEA and ZAL and their primary metabolites, zearalenol, zearalanone, and their conjugated glucuronides, for rats and for human subjects. The PBTK modeling study explicitly simulates critical metabolic pathways in the gastrointestinal and hepatic systems. Metabolic events such as dehydrogenation and glucuronidation of the chemicals, which have direct effects on the accumulation and elimination of the toxic compounds, have been quantified. The PBTK model considers urinary and fecal excretion and biliary recirculation and compares the predicted biomarkers of blood, urinary and fecal concentrations with published in vivo measurements in rats and human subjects. Additionally, the toxicokinetic model has been coupled with a novel probabilistic dietary exposure model and applied to the Jersey Girl Study (JGS), which involved measurement of mycoestrogens as urinary biomarkers, in a cohort of young girls in New Jersey, USA. A probabilistic exposure characterization for the study population has been conducted and the predicted urinary concentrations have been compared to measurements considering inter-individual physiological and dietary variability. The in vivo measurements from the JGS fall within the high and low predicted distributions of biomarker values corresponding to dietary exposure estimates calculated by the probabilistic modeling system. The work described here is the first of its kind to present a comprehensive framework developing estimates of potential exposures to mycotoxins and linking them with biologically relevant doses and biomarker measurements, including a systematic characterization of uncertainties in exposure and dose estimation for a vulnerable population.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4256163/

Record 1279

pmcid: PMC4208259

pmid: 25346921

doi: 10.6065/apem.2014.19.3.159

title: Prepubertal unilateral gynecomastia in the absence of endocrine abnormalities.

authors

- Kang M
- Lee CJ
- Hwang IT
- Lee K
- Kang MJ

journal: Annals of pediatric endocrinology & metabolism

journalAbbr: Ann Pediatr Endocrinol Metab

year: 2014

pubdate: 2014 Sep

volume: 19

issue: 3

pages: 159-63

abstract: Gynecomastia is defined as the excessive development of breast tissue in males, which can occur as unilateral or bilateral. Although the overall prevalence of gynecomastia is 40%-65%, the prepubertal unilateral gynecomastia in the absence of endocrine abnormalities is unusual, with only a few case reports in literature. Idiopathic gynecomastia in prepuberty is benign but a diagnosis of exclusion. We here report two cases of healthy prepubertal boys aged 8.8 and 9.6 years old, respectively, presented with painful palpable mass in their unilateral breast. Breast tissue with glandular proliferation was confirmed on ultrasonography. Serum levels of estradiol, testosterone, and other laboratory findings were within normal range. They seem to have the prepubertal idiopathic gynecomastia but further follow-up to see their progression is needed.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4208259/

Record 1280

pmcid: PMC4293661

pmid: 25541897

doi: 10.4274/Jcrpe.1477

title: Prepubertal unilateral gynecomastia: report of 2 cases.

authors

- Demirbilek H
- Bacak G
- Baran RT
- Avcı Y
- Baran A
- Keleş A
- Özbek MN
- Alanay Y
- Hussain K

journal: Journal of clinical research in pediatric endocrinology

journalAbbr: J Clin Res Pediatr Endocrinol

year: 2014

pubdate: 2014 Dec

volume: 6

issue: 4

pages: 250-3

abstract: Prepubertal unilateral gynecomastia is an extremely rare condition. At present, its etiology and management strategy are not well known. Two unrelated prepubertal boys of ages 8 and 9 who presented with complaints of unilateral enlargement of breast tissue are reported. Physical examination, biochemical, hormonal and oncologic work-up findings were normal. Both patients were treated with peripheral liposuction successfully. Histopathological and immunohistochemical examinations showed benign fibroglandular gynecomastia and intensive (3+) estrogen receptor expression in 100% of periductal epithelial cells. Although an extremely rare and generally benign condition, patients with prepubertal unilateral gynecomastia should have a full endocrine and oncologic work-up.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4293661/

Record 1281

pmcid: PMC4278570

pmid: 24510381

doi: 10.1002/cncr.28550

title: Primary tumor resection as a component of multimodality treatment may improve local control and survival in patients with stage IV inflammatory breast cancer.

authors

- Akay CL
- Ueno NT

- Chisholm GB
- Hortobagyi GN
- Woodward WA
- Alvarez RH
- Bedrosian I
- Kuerer HM
- Hunt KK
- Huo L
- Babiera GV

journal: Cancer

journalAbbr: Cancer

year: 2014

pubdate: 2014 May 1

volume: 120

issue: 9

pages: 1319-28

abstract: BACKGROUND: To the authors' knowledge, the benefit of primary tumor resection among patients with metastatic inflammatory breast cancer (IBC) is unknown. METHODS: The authors reviewed 172 cases of metastatic IBC. All patients received chemotherapy with or without radiotherapy and/or surgery. Patients were classified as responders or nonresponders to chemotherapy. The 5-year overall survival (OS) and distant progression-free survival (DPFS) and local control at the time of last follow-up were evaluated. RESULTS: A total of 79 patients (46%) underwent surgery. OS and DPFS were better among patients treated with surgery versus no surgery (47% vs 10%, respectively [P<.0001] and 30% vs 3%, respectively [P<.0001]). Surgery plus radiotherapy was associated with better survival compared with treatment with surgery or radiotherapy alone (OS rate: 50% vs 25% vs 14%, respectively; DPFS rate: 32% vs 18% vs 15%, respectively [P<.0001 for both]). Surgery was associated with better survival for both responders (OS rate for surgery vs no surgery: 49% vs 23% [P<.0001] and DPFS rate for surgery vs no surgery: 31% vs 8% [P<.0001]) and nonresponders (OS rate for surgery vs no surgery: 40% vs 6% [P<.0001] and DPFS rate for surgery vs no surgery: 30% vs 0% [P<.0001]). On multivariate analysis, treatment with surgery plus radiotherapy and response to chemotherapy were found to be significant predictors of better OS and DPFS. Local control at the time of last follow-up was 4-fold more likely in patients who underwent surgery with or without radiotherapy compared with patients who received chemotherapy alone (81% vs 18%; P<.0001). Surgery and response to chemotherapy independently predicted local control on multivariate analysis. CONCLUSIONS: The results of the current study demonstrate that for select patients with metastatic IBC, multimodality treatment including primary tumor resection may result in better local control and survival. However, a randomized trial is needed to validate these findings.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4278570/

Record 1282

pmcid: PMC3995375

pmid: 24803884

doi: 10.1159/000358751

title: Psychological impact of male breast disorders: literature review and survey results.

authors

- Kipling M
- Ralph JE
- Callanan K

journal: Breast care (Basel, Switzerland)

journalAbbr: Breast Care (Basel)

year: 2014

pubdate: 2014 Feb

volume: 9

issue: 1

pages: 29-33

abstract: BACKGROUND: Referrals of men to breast assessment clinics are increasing. While most of the men will have benign disease, some of them will have breast cancer. Whichever pathology they have, men should be offered a service tailored to their needs, rather than being 'shoehorned' into a service designed to care for women. This paper explores the psychological impact on men of their condition and of attending a breast assessment clinic. METHODS: The literature regarding male experience of breast problems is reviewed, and screening for psychological morbidity is discussed. Results of a survey regarding an all-male breast assessment clinic are reported, with a plan for future research. RESULTS: Many of the 78 men surveyed described negative feelings relating to their condition although they did not want to be seen in an all-male breast assessment clinic if that meant a longer wait. Men reported feelings of anxiety, embarrassment, emasculation and even depression regarding their condition. CONCLUSIONS: Men are distressed by gynaecomastia and need psychological support for any breast-related presentation. More formalised research into this area is needed, although the men's distress does not translate into the desire to attend an all-male assessment clinic if this means a longer wait before being seen.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3995375/

Record 1283

pmcid: PMC3979124

pmid: 24433463

doi: 10.1186/ar4441

title: Review of pharmacological therapies in fibromyalgia syndrome.

authors

- Häuser W
- Walitt B
- Fitzcharles MA
- Sommer C

journal: Arthritis research & therapy

journalAbbr: Arthritis Res Ther

year: 2014

pubdate: 2014 Jan 17

volume: 16

issue: 1

pages: 201

abstract: This review addresses the current status of drug therapy for the management of fibromyalgia syndrome (FMS) and is based on interdisciplinary FMS management guidelines, meta-analyses of drug trial data, and observational studies. In the absence of a single gold-standard medication, patients are treated with a variety of drugs from different categories, often with limited evidence. Drug therapy is not mandatory for the management of FMS. Pregabalin, duloxetine, milnacipran, and amitriptyline are the current first-line prescribed agents but have had a mostly modest effect. With only a minority of patients expected to experience substantial

benefit, most will discontinue therapy because of either a lack of efficacy or tolerability problems. Many drug treatments have undergone limited study and have had negative results. It is unlikely that these failed pilot trials will undergo future study. However, medications, though imperfect, will continue to be a component of treatment strategy for these patients. Both the potential for medication therapy to relieve symptoms and the potential to cause harm should be carefully considered in their administration.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3979124/

Record 1284

pmcid: PMC4134643

pmid: 25136214

doi: 10.4103/0974-2077.138354

title: Role of Combined Circumareolar Skin Excision and Liposuction in Management of High grade Gynaecomastia.

authors

- Sarkar A
- Bain J
- Bhattacharya D
- Sawarappa R
- Munian K
- Dutta G
- Naiyer GJ
- Ahmad S

journal: Journal of cutaneous and aesthetic surgery

journalAbbr: J Cutan Aesthet Surg

year: 2014

pubdate: 2014 Apr

volume: 7

issue: 2

pages: 112-6

abstract: INTRODUCTION: High-grade gynaecomastia (Simon IIb and III) has tissue excess (skin excess, enlarged areola, and displaced nipple), which is best managed surgically; however, results of conventional breast reduction surgeries and liposuction is not very good. Aim of our study was to describe a combined technique to manage these problems to produce a good result. MATERIAL AND METHOD: This was a 2-year study among 12 patients of high grade gynaecomastia. Clinical and laboratory findings were normal. Pre-operatively in standing position, diameter of breast and areola, position of nipple, and amount of skin excess were marked. Under general anaesthesia, tumescent infiltration, circumareolar de-epithelisation of skin excess, and liposuction was completed. Redundant portion of the breast was sharply dissected and pulled out. Areola was fixed over pectoralis fascia at mid humerus level, just medial to the mid-clavicular line. Outer borders of the de-epithelised area were apposed by the purse-string effect of a subdermal suture, and further apposed by few half buried horizontal mattress sutures. Drains for 24 hour and compressive dressings for 6 weeks were used. RESULT: Mean age of presentation was 25.8 year; emotional discomfort was the chief complaint. Among 12 patients, 10 patients had bilateral gynaecomastia and 8 patients had enlarged and displaced nipple-areola complex. Average hospital stay was 2.41 days and recoveries were usually uneventful. CONCLUSION: The problem of tissue excess and tissue displacement in high grade gynaecomastia can be well managed by this combined circumareolar skin reduction and liposuction technique to achieve a scar-less flat male chest.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4134643/

Record 1285

pmcid: PMC5785096

pmid: 24041956

doi: 10.1016/j.brachy.2013.08.002

title: RTOG 95-17, a Phase II trial to evaluate brachytherapy as the sole method of radiation therapy for Stage I and II breast carcinoma--year-5 toxicity and cosmesis.

authors

- Rabinovitch R
- Winter K
- Kuske R
- Bolton J
- Arthur D
- Scroggins T
- Vicini F
- McCormick B
- White J

journal: Brachytherapy

journalAbbr: Brachytherapy

year: 2014

pubdate: 2014 Jan-Feb

volume: 13

issue: 1

pages: 17-22

abstract: PURPOSE: Radiation Therapy Oncology Group (RTOG) 95-17, a Phase II trial to evaluate multicatheter brachytherapy (mCathBrachy) as the sole method of radiation therapy for Stage I-II breast cancer (BrCa), was the first cooperative group trial in North America to evaluate accelerated partial breast irradiation (APBI) and include patient-reported outcomes (PROs). This report presents the year-5 toxicity and cosmesis data. METHODS AND MATERIALS: After lumpectomy and axillary dissection for invasive BrCa (tumor size <3cm with zero to three positive lymph nodes), 100 patients (pts), 98 evaluable, were treated (txed) with mCathBrachy from 1997 to 2000 with 34Gy administered twice daily in 10 high-dose-rate fractions or 45Gy in 3.5-6 days as a low-dose-rate implant to 1-2cm beyond the lumpectomy bed. The PROs and physician-reported outcomes of toxicity, cosmesis, and tx satisfaction at year-5 are reported here, defined as data submitted 54-78 months after tx. RESULTS: Grade (G) 1-2 skin toxicity developed in 78% of the pts and G3 in 13% (no G4). The tx effects included skin dimpling/indentation (37%), fibrosis (45%), telangiectasias (45%), skin catheter marks (54%), and symptomatic fat necrosis (15%). Breast asymmetry was reported in 73%. Rates of excellent-to-good cosmesis were similar between PROs (66%) and radiation oncologists (68%). The PROs of tx satisfaction at year-5 was 75%. CONCLUSIONS: RTOG 95-17 documents the year-5 skin toxicity and tx effects of mCathBrachy APBI, which are associated with PROs of good-to-excellent cosmesis and high tx satisfaction. This emphasizes the importance of PROs when assessing BrCa tx. National Surgical Adjuvant Breast and Bowel Project B39/RTOG 0413 will allow for definitive comparisons between APBI and whole breast radiation therapy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5785096/

Record 1286

pmcid: PMC4160543

pmid: 25178390

doi: 10.1186/1471-2334-14-474

title: Safety of poly-L-lactic acid (New-Fill®) in the treatment of facial lipoatrophy: a large observational study among HIV-positive patients.

authors

- Duracinsky M
- Leclercq P
- Herrmann S
- Christen MO
- Dolivo M
- Goujard C
- Chassany O

journal: BMC infectious diseases

journalAbbr: BMC Infect Dis

year: 2014

pubdate: 2014 Sep 1

volume: 14

issue:

pages: 474

abstract: BACKGROUND: Facial lipoatrophy is a frequently reported condition associated with use of antiretroviral (ARV) drugs. Poly-L-lactic acid (PLLA) acid has been used to correct facial lipoatrophy in people with HIV since 2004 both in Europe and the United States. The objective of this study was to establish, in real life conditions and in a large sample, the safety of PLLA (New Fill®, Valeant US, Sinclair Pharma Paris, France) to correct facial lipoatrophy among HIV-positive patients. METHODS: A longitudinal study was conducted between 2005 and 2008 in France. Data from 4,112 treatment courses (n = 4,112 patients) and 15,665 injections sessions (1 to 5 injection sessions per treatment course) were gathered by 200 physicians trained in the use of PLLA. RESULTS: The average age of patients (88.3% males) treated for lipoatrophy was 47.1 ± 8.1 years (Mean ± SD); 91.2% of patients had been receiving ARV treatment for 10.9 (±4.2) years; CD4 T-cell count was 535 ± 266 cells/mm3. The duration of facial lipoatrophy was 5 ± 2.8 years and the severity was such that 47.3% of patients required five injection sessions of PLLA and 81.9% of the sessions required two vials of the preparation. The final visit, scheduled two months after the last injection session, was attended by 66.0% of patients (n = 2,713). 48 treatment courses (2.8%) were discontinued due to adverse events (AEs). The overall incidence of AEs per course was 18.8%. Immediate AEs, bleeding (3.4%), bruising (2.3%), pain (2.0%), redness at injection site (1.6%), and swelling of the face (0.7%), occurred in 15.4% of courses and 7.0% of sessions (usually during the first session). Non-immediate AEs, mainly nodules (5.7%), inflammation (0.7%), granuloma (0.3%), discolouration (0.2%), and skin hypertrophy (0.1%), occurred in 6.7% of courses. Non-immediate AEs occurred within a time ranging from 21 days (inflammation) to 101 days (granuloma) and all but three of the 13 cases of granuloma resolved. Product efficacy was rated satisfactory by 95% of the patients and physicians. CONCLUSIONS: This study demonstrated, in real-life conditions and on a large sample, that PLLA injections were feasible, efficient, and safe when performed by trained physicians.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4160543/

Record 1287

pmcid: PMC4255892

pmid: 25506532

doi: 10.1097/GOX.0000000000000212

title: Soft, Brown Rupture: Clinical Signs and Symptoms Associated with Ruptured PIP Breast Implants.

authors

- Godwin Y
- Duncan RT
- Feig C
- Reintals M
- Hill S

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2014

pubdate: 2014 Nov

volume: 2

issue: 11

pages: e249

abstract: BACKGROUND: Preoperative signs and symptoms of patients with Poly Implant Prothese (PIP) implants could be predictive of device failure. Based on clinical observation and intraoperative findings 4 hypotheses were raised: (1) Preoperative clinical signs including acquired asymmetry, breast enlargement, fullness of the lower pole, decreased mound projection, and change in breast consistency could be indicative of implant rupture. (2) Device failure correlates with a low preoperative Baker grade of capsule. (3) Brown-stained implants are more prone to implant failure. (4) The brown gel could be indicative of iodine ingression through a substandard elastomer shell. METHODS: Preoperative clinical signs were compared with intraoperative findings for 27 patients undergoing PIP implant explantation. RESULTS: Acquired asymmetry (P = 0.0003), breast enlargement (P = 0.0002), fuller lower pole (P < 0.0001), and loss of lateral projection (P < 0.0001) were all significantly predictive of device failure. Capsule Baker grade was lower preoperatively for ruptured implants. The lack of palpable and visible preoperative capsular contracture could be secondary to the elastic nature of the capsular tissue found. Brown implants failed significantly more often than white implants. Analysis of brown gel revealed the presence of iodine, suggesting povidone iodine ingression at implantation. CONCLUSIONS: Preoperative signs can be predictive of PIP implant failure. Brown-stained implants are more prone to rupture. The presence of iodine in the gel suggests unacceptable permeability of the shell early in the implant's life span. A noninvasive screening test to detect brown implants in situ could help identify implants at risk of failure in those who elect to keep their implants.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4255892/

Record 1288

pmcid: PMC4260272

pmid: 25538910

doi: 10.4103/2277-9175.145696

title: Some side effects and effcts on physical activity of second-generation antipsychotics: A study in children and adolescents.

authors

- Arman S
- Sadeghye T
- Bidaki R

journal: Advanced biomedical research

journalAbbr: Adv Biomed Res

year: 2014

pubdate: 2014

volume: 3

issue:

pages: 224

abstract: BACKGROUND: This study was designed to investigate the metabolic adverse effects (AEs) of second-generation antipsychotics (SGAs) and their relationship with physical activity and non-metabolic AE in children and adolescents. MATERIALS AND METHODS: After exclusion of patients with metabolic syndrome, 62 patients (34 children, 28 adolescents) of both genders who were candidates for SGA therapy were selected. Metabolic parameters included fasting blood glucose (FBG), triglyceride (TG), blood pressure (BP), and waist circumference (WC); non-metabolic AEs and physical activity were evaluated at baseline, 1 month, and 3 months after starting the treatment. RESULTS: Mean of post-treatment FBG and TG were significantly higher than the baseline values (P < 0.0001). Compared to the baseline value, significantly more patients developed abnormally high (AbH) FBG at the end point (P = 0.02). There was no significant difference in the frequency of patients with AbH-FBG either at the baseline or at the end point (P > 0.05). The frequency of patients with AbH-TG at the end point was not significantly higher than those with baseline AbH-TG (P = 0.10). Although no patient was obese at baseline, 11 (18%) patients developed abdominal obesity at the end point (P < 0.0001). There was no significant difference in the frequency of non-metabolic AE (P > 0.05). There was no significant correlation between metabolic and non-metabolic AE (P > 0.05). Frequency of inactive patients was significantly more than the baseline value (P-0.008), and abdominal obesity was significantly more prevalent in less active participants (P = 0.03). CONCLUSION: The present study showed the AE of SGA on FBG and TG, but no effect on BP and WC. We also found that children are more prone to develop abnormally high FBG.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4260272/

Record 1289

pmcid: PMC4089122

pmid: 25024978

doi: 10.4103/2249-4847.134701

title: Sonographic features of physiologic neonatal breast enlargement.

authors

- Jawahar A
- Vade A

journal: Journal of clinical neonatology

journalAbbr: J Clin Neonatol

year: 2014

pubdate: 2014 Apr

volume: 3

issue: 2

pages: 106-8

abstract: The varying presentations of neonatal breast enlargement on imaging have been underreported in the literature. Our case report profiles a 3-week-old female patient who presented with a history of left breast enlargement with redness and tenderness for 2 days, who was clinically diagnosed and managed for neonatal mastitis, which was actually a neonatal breast enlargement with adjacent cellulitis. Awareness that physiologic neonatal breast enlargement can be associated with adjacent cellulitis without mastitis can prevent unnecessary hospitalization and treatment with parenteral antibiotics.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4089122/

Record 1290

pmcid: PMC4230663

pmid: 25387499

doi: 10.1590/abd1806-4841.20142873

title: Surgical treatment of axillary hyperhidrosis by suction-curettage of sweat glands.

authors

- Rezende RM
- Luz FB

journal: Anais brasileiros de dermatologia

journalAbbr: An Bras Dermatol

year: 2014

pubdate: 2014 Nov-Dec

volume: 89

issue: 6

pages: 940-54

abstract: UNLABELLED: Suction curettage is a dermatologic surgery technique for the treatment of axillary hyperhidrosis, which is becoming more popular. OBJECTIVE: The purpose of this study is to describe the current technique of removal of axillary sweat glands, and evaluate its efficacy and safety. CONCLUSION: Suction-curettage of sweat glands is a minimally invasive surgical technique that is easy to perform, safe, has high rates of success and relatively few side-effects. It is generally well tolerated by patients and requires shorter time away from daily activities, when compared with other surgical modalities.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4230663/

Record 1291

pmcid: PMC3985532

pmid: 24708809

doi: 10.1186/1477-7819-12-88

title: Synchronous invasive ductal carcinoma and intravascular large B-cell lymphoma of the breast: a case report and review of the literature.

authors

- Ho CW
- Mantoo S
- Lim CH
- Wong CY

journal: World journal of surgical oncology

journalAbbr: World J Surg Oncol

year: 2014

pubdate: 2014 Apr 8

volume: 12

issue:

pages: 88

abstract: Primary breast lymphomas (PBLs) represent less than 1% of all breast malignancies. Intravascular large B-cell lymphoma (ILBCL) is a rare, aggressive form of extranodal lymphoma. Breast involvement has only

been described in the literature once previously. ILBCL is characterized by the proliferation of tumour cells within the lumen of small vessels of involved organs, resulting in their eventual occlusion. Clinical features are often vague, diagnosis is difficult and delayed, and prognosis is usually poor. We report the first ever case of synchronous ILBCL and invasive ductal carcinoma (IDC) of the breast in a patient presenting with pyrexia of unknown origin and altered mental status who underwent modified radical mastectomy and subsequent chemotherapy, and review the literature regarding intravascular large B-cell lymphoma, PBLs and synchronous carcinomas and lymphomas of the breast.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3985532/

Record 1292

pmcid: PMC4135458

pmid: 25249706

doi: 10.1179/2050854914Y.0000000027

title: The breast cancer epidemic: 10 facts.

authors

- Schneider AP 2nd
- Zainer CM
- Kubat CK
- Mullen NK
- Windisch AK

journal: The Linacre quarterly

journalAbbr: Linacre Q

year: 2014

pubdate: 2014 Aug

volume: 81

issue: 3

pages: 244-77

abstract: Breast cancer, affecting one in eight American women, is a modern epidemic. The increasing frequency of breast cancer is widely recognized. However, the wealth of compelling epidemiological data on its prevention is generally not available, and as a consequence, is largely unknown to the public. The purpose of this report is to review the epidemiological evidence of preventable causes of breast cancer. [Table: see text].

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4135458/

Record 1293

pmcid: PMC4022988

pmid: 24677189

doi: 10.1007/s40263-014-0157-3

title: The effects of novel and newly approved antipsychotics on serum prolactin levels: a comprehensive review.

authors

- Peuskens J
- Pani L
- Detraux J
- De Hert M

journal: CNS drugs

journalAbbr: CNS Drugs

year: 2014

pubdate: 2014 May

volume: 28

issue: 5

pages: 421-53

abstract: Since the 1970s, clinicians have increasingly become more familiar with hyperprolactinemia (HPRL) as a common adverse effect of antipsychotic medication, which remains the cornerstone of pharmacological treatment for patients with schizophrenia. Although treatment with second-generation antipsychotics (SGAs) as a group is, compared with use of the first-generation antipsychotics, associated with lower prolactin (PRL) plasma levels, the detailed effects on plasma PRL levels for each of these compounds in reports often remain incomplete or inaccurate. Moreover, at this moment, no review has been published about the effect of the newly approved antipsychotics asenapine, iloperidone and lurasidone on PRL levels. The objective of this review is to describe PRL physiology; PRL measurement; diagnosis, causes, consequences and mechanisms of HPRL; incidence figures of (new-onset) HPRL with SGAs and newly approved antipsychotics in adolescent and adult patients; and revisit lingering questions regarding this hormone. A literature search, using the MEDLINE database (1966-December 2013), was conducted to identify relevant publications to report on the state of the art of HPRL and to summarize the available evidence with respect to the propensity of the SGAs and the newly approved antipsychotics to elevate PRL levels. Our review shows that although HPRL usually is defined as a sustained level of PRL above the laboratory upper limit of normal, limit values show some degree of variability in clinical reports, making the interpretation and comparison of data across studies difficult. Moreover, many reports do not provide much or any data detailing the measurement of PRL. Although the highest rates of HPRL are consistently reported in association with amisulpride, risperidone and paliperidone, while aripiprazole and quetiapine have the most favorable profile with respect to this outcome, all SGAs can induce PRL elevations, especially at the beginning of treatment, and have the potential to cause new-onset HPRL. Considering the PRL-elevating propensity of the newly approved antipsychotics, evidence seems to indicate these agents have a PRL profile comparable to that of clozapine (asenapine and iloperidone), ziprasidone and olanzapine (lurasidone). PRL elevations with antipsychotic medication generally are dose dependant. However, antipsychotics having a high potential for PRL elevation (amisulpride, risperidone and paliperidone) can have a profound impact on PRL levels even at relatively low doses, while PRL levels with antipsychotics having a minimal effect on PRL, in most cases, can remain unchanged (quetiapine) or reduce (aripiprazole) over all dosages. Although tolerance and decreases in PRL values after long-term administration of PRL-elevating antipsychotics can occur, the elevations, in most cases, remain above the upper limit of normal. PRL profiles of antipsychotics in children and adolescents seem to be the same as in adults. The hyperprolactinemic effects of antipsychotic medication are mostly correlated with their affinity for dopamine D2 receptors at the level of the anterior pituitary lactotrophs (and probably other neurotransmitter mechanisms) and their blood-brain barrier penetrating capability. Even though antipsychotics are the most common cause of pharmacologically induced HPRL, recent research has shown that HPRL can be pre-existing in a substantial portion of antipsychotic-naïve patients with first-episode psychosis or at-risk mental state.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4022988/

Record 1294

pmcid: PMC4203468

pmid: 25332722

doi: 10.1186/1746-4358-9-17

title: The impact of cosmetic breast implants on breastfeeding: a systematic review and meta-analysis.

authors

- Schiff M

- Algert CS
- Ampt A
- Sywak MS
- Roberts CL

journal: International breastfeeding journal

journalAbbr: Int Breastfeed J

year: 2014

pubdate: 2014

volume: 9

issue:

pages: 17

abstract: BACKGROUND: Cosmetic breast augmentation (breast implants) is one of the most common plastic surgery procedures worldwide and uptake in high income countries has increased in the last two decades. Women need information about all associated outcomes in order to make an informed decision regarding whether to undergo cosmetic breast surgery. We conducted a systematic review to assess breastfeeding outcomes among women with breast implants compared to women without. METHODS: A systematic literature search of Medline, Pubmed, CINAHL and Embase databases was conducted using the earliest inclusive dates through December 2013. Eligible studies included comparative studies that reported breastfeeding outcomes (any breastfeeding, and among women who breastfed, exclusive breastfeeding) for women with and without breast implants. Pairs of reviewers extracted descriptive data, study quality, and outcomes. Rate ratios (RR) and 95% confidence intervals (CI) were pooled across studies using the random-effects model. The Newcastle-Ottawa scale (NOS) was used to critically appraise study quality, and the National Health and Medical Research Council Level of Evidence Scale to rank the level of the evidence. This systematic review has been registered with the international prospective register of systematic reviews (PROSPERO): CRD42014009074. RESULTS: Three small, observational studies met the inclusion criteria. The quality of the studies was fair (NOS 4-6) and the level of evidence was low (III-2 - III-3). There was no significant difference in attempted breastfeeding (one study, RR 0.94, 95% CI 0.76, 1.17). However, among women who breastfed, all three studies reported a reduced likelihood of exclusive breastfeeding amongst women with breast implants with a pooled rate ratio of 0.60 (95% CI 0.40, 0.90). CONCLUSIONS: This systematic review and meta-analysis suggests that women with breast implants who breastfeed were less likely to exclusively feed their infants with breast milk compared to women without breast implants.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4203468/

Record 1295

pmcid: PMC5060692

pmid: 22032636

doi: 10.1111/j.1369-7625.2011.00734.x

title: To accept, or not to accept, that is the question: citizen reactions to rationing.

authors

- Broqvist M
- Garpenby P

journal: Health expectations : an international journal of public participation in health care and health policy

journalAbbr: Health Expect

year: 2014

pubdate: 2014 Feb

volume: 17

issue: 1

pages: 82-92

abstract: BACKGROUND: The publicly financed health service in Sweden has come under increasing pressure, forcing policy makers to consider restrictions. OBJECTIVE: To describe different perceptions of rationing, in particular, what citizens themselves believe influences their acceptance of having to stand aside for others in a public health service. DESIGN: Qualitative interviews, analysed by phenomenography, describing perceptions by different categories. SETTING AND PARTICIPANTS: Purposeful sample of 14 Swedish citizens, based on demographic criteria and attitudes towards allocation in health care. RESULTS: Participants expressed high awareness of limitations in public resources and the necessity of rationing. Acceptance of rationing could increase or decrease, depending on one's (i) awareness that healthcare resources are limited, (ii) endorsement of universal health care, (iii) knowledge and acceptance of the principles guiding rationing and (iv) knowledge about alternatives to public health services. CONCLUSIONS: This study suggests that decision makers should be more explicit in describing the dilemma of resource limitations in a publicly funded healthcare system. Openness enables citizens to gain the insight to make informed decisions, i.e. to use public services or to 'opt out' of the public sector solution if they consider rationing decisions unacceptable.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5060692/

Record 1296

pmcid: PMC4162655

pmid: 25264451

doi: 10.1586/17446651.2014.926810

title: Understanding the pathological manifestations of aromatase excess syndrome: lessons for clinical diagnosis.

authors

- Shozu M
- Fukami M
- Ogata T

journal: Expert review of endocrinology & metabolism

journalAbbr: Expert Rev Endocrinol Metab

year: 2014

pubdate: 2014 Jul

volume: 9

issue: 4

pages: 397-409

abstract: Aromatase excess syndrome is characterized by pre- or peripubertal onset of gynecomastia due to estrogen excess because of a gain-of-function mutation in the aromatase gene (CYP19A1). Subchromosomal recombination events including duplication, deletion, and inversion has been identified. The latter two recombinations recruit novel promoters for CYP19A1 through a unique mechanism. Gynecomastia continues for life, and although the general condition is well preserved, it may cause psychological issues. Minor symptoms (variably advanced bone age and short adult height), if present, are exclusively because of estrogen excess. Serum estradiol levels are elevated in 48% of affected males, but are not necessarily useful for diagnosis. Molecular analysis of CYP19A1 mutations is mandatory to confirm aromatase excess syndrome diagnosis. Furthermore, the use of an aromatase inhibitor can ameliorate gynecomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4162655/

Record 1297

pmcid: PMC3683948

pmid: 24415964

doi: 10.1159/000350774

title: A Brief Overview of the WHO Classification of Breast Tumors, 4th Edition, Focusing on Issues and Updates from the 3rd Edition.

authors

- Sinn HP
- Kreipe H

journal: Breast care (Basel, Switzerland)

journalAbbr: Breast Care (Basel)

year: 2013

pubdate: 2013 May

volume: 8

issue: 2

pages: 149-54

abstract: The WHO Classification of Tumors of the Breast, 4th edition, is an update to the 3rd edition that was published in 2003, and covers all neoplastic and preneoplastic lesions of the breast. Changes to the 4th edition include new aspects and changes to the terminology that reflect our present-day knowledge of these lesions. Definitions for histopathological diagnosis are complemented by a description of clinical features, epidemiology, macroscopy, genetics, and prognostic and predictive features. In this review of the WHO classification, we have focused on invasive carcinomas, precursor lesions, and some benign epithelial proliferations.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3683948/

Record 1298

pmcid: PMC3754409

pmid: 24040602

doi:

title: A case of van wyk-grumbach syndrome.

authors

- Iqbal MZ
- Saleem M
- Shahzad ZA

journal: APSP journal of case reports

journalAbbr: APSP J Case Rep

year: 2013

pubdate: 2013

volume: 4

issue: 2

pages: 24

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3754409/

Record 1299

pmcid: PMC3872162

pmid: 24385983

doi: 10.1155/2013/591012

title: A network investigation on idiopathic hypogonadotropic hypogonadism in china.

authors

- Zhao W
- Ye H
- Zhao X
- Zhang Z
- Sun S
- Jiang Y
- He M
- Xu C
- Hu R
- Li Y

journal: International journal of endocrinology

journalAbbr: Int J Endocrinol

year: 2013

pubdate: 2013

volume: 2013

issue:

pages: 591012

abstract: Idiopathic hypogonadotropic hypogonadism (IHH) is a rare condition in which puberty does not take place naturally. We aimed to develop and follow an internet-based cohort and to improve our understanding of the disease. We established an internet-based questionnaire survey. A total of 74 male IHH patients were recruited from the Chinese largest IHH network social group. The clinical symptoms before treatment mainly included small testis, underdeveloped secondary sexual characteristics, and sexual dysfunction. After treatment, the penis length, testicular volume, external genital organ development, pubic hair, beard, laryngeal prominence, erection, and spermatorrhea were improved significantly ($P < 0.001$). 18.9% of the patients completed fertility; however, more than half of the patients still complained of poor happiness and low physical strength. In addition, improvements in penis and pubic hair development, testosterone normalization and the physical strength in IHH patients who received gonadotropin and androgen replacement therapy were better than in those who received single gonadotropin therapy ($P < 0.05$ for all). In conclusion, disease-specific network investigation can be used as an alternative method of medical research for rare diseases. The results of our cross-sectional study showed the effectiveness of hormone replacement therapy for IHH and implied that gonadotropin and androgen replacement therapy may be superior to gonadotropin treatment alone.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3872162/

Record 1300

pmcid: PMC3840809

pmid: 24294485

doi: 10.1177/2045125313490305

title: A new logical insight and putative mechanism behind fluoxetine-induced amenorrhea, hyperprolactinemia and galactorrhea in a case series.

authors

- Mondal S
- Saha I
- Das S
- Ganguly A
- Das D
- Tripathi SK

journal: Therapeutic advances in psychopharmacology

journalAbbr: Ther Adv Psychopharmacol

year: 2013

pubdate: 2013 Dec

volume: 3

issue: 6

pages: 322-34

abstract: With the exception of fluoxetine, all selective serotonin reuptake inhibitors (SSRIs) commonly cause hyperprolactinemia through presynaptic mechanisms indirectly via 5-hydroxytryptamine (5-HT)-mediated inhibition of tuberoinfundibular dopaminergic neurons. However, there is little insight regarding the mechanisms by which fluoxetine causes hyperprolactinemia via the postsynaptic pathway. In this text, analysis of five spontaneously reported clinical cases of hyperprolactinemia resulting in overt symptoms of amenorrhea with or without galactorrhea, were scrupulously analyzed after meticulously correlating relevant literature and an attempt was made to explore the putative postsynaptic pathway of fluoxetine inducing hyperprolactinemia. Hypothetically, serotonin regulates prolactin release either by increasing oxytocin (OT) level via direct stimulation of vasoactitive intestinal protein (VIP) or indirectly through stimulation of GABAergic neurons. The pharmacodynamic exception and pharmacokinetic aspect of fluoxetine are highlighted to address the regulation of prolactin release via serotonergic pathway, either directly through stimulation of prolactin releasing factors (PRFs) VIP and OT via 5-HT2A receptors predominantly on PVN (neurosecretory magnocellular cell) or through induction of 5-HT1A-mediated direct and indirect GABAergic actions. Prospective molecular and pharmacogenetic studies are warranted to visualize how fluoxetine regulate neuroendocrine system and cause adverse consequences, which in turn may explore new ways of approach of drug development by targeting the respective metabolic pathways to mitigate these adverse impacts.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3840809/

Record 1301

pmcid: PMC3653687

pmid: 23547564

doi: 10.1186/1745-6215-14-91

title: A pilot study of the effect of spironolactone therapy on exercise capacity and endothelial dysfunction in pulmonary arterial hypertension: study protocol for a randomized controlled trial.

authors

- Elinoff JM
- Rame JE
- Forfia PR
- Hall MK
- Sun J
- Gharib AM

- Abd-Elmoniem K
- Graninger G
- Harper B
- Danner RL
- Solomon MA

journal: Trials

journalAbbr: Trials

year: 2013

pubdate: 2013 Apr 2

volume: 14

issue:

pages: 91

abstract: BACKGROUND: Pulmonary arterial hypertension is a rare disorder associated with poor survival. Endothelial dysfunction plays a central role in the pathogenesis and progression of pulmonary arterial hypertension. Inflammation appears to drive this dysfunctional endothelial phenotype, propagating cycles of injury and repair in genetically susceptible patients with idiopathic and disease-associated pulmonary arterial hypertension. Therapy targeting pulmonary vascular inflammation to interrupt cycles of injury and repair and thereby delay or prevent right ventricular failure and death has not been tested. Spironolactone, a mineralocorticoid and androgen receptor antagonist, has been shown to improve endothelial function and reduce inflammation. Current management of patients with pulmonary arterial hypertension and symptoms of right heart failure includes use of mineralocorticoid receptor antagonists for their diuretic and natriuretic effects. We hypothesize that initiating spironolactone therapy at an earlier stage of disease in patients with pulmonary arterial hypertension could provide additional benefits through anti-inflammatory effects and improvements in pulmonary vascular function. METHODS/DESIGN: Seventy patients with pulmonary arterial hypertension without clinical evidence of right ventricular failure will be enrolled in a randomized, double-blinded, placebo-controlled trial to investigate the effect of early treatment with spironolactone on exercise capacity, clinical worsening and vascular inflammation in vivo. Our primary endpoint is change in placebo-corrected 6-minute walk distance at 24 weeks and the incidence of clinical worsening in the spironolactone group compared to placebo. At a two-sided alpha level of 0.05, we will have at least 84% power to detect an effect size (group mean difference divided by standard deviation) of 0.9 for the difference in the change of 6-minute walk distance from baseline between the two groups. Secondary endpoints include the effect of spironolactone on the change in placebo-corrected maximal oxygen consumption; plasma markers of vascular inflammation and peripheral blood mononuclear cell gene expression profiles; sympathetic nervous system activation, renin-angiotensin-aldosterone system activation and sex hormone metabolism; and right ventricular structure and function using echocardiography and novel high-resolution magnetic resonance imaging-based techniques. Safety and tolerability of spironolactone will be assessed with periodic monitoring for hyperkalemia and renal insufficiency as well as the incidence of drug discontinuation for untoward effects. TRIAL REGISTRATION: ClinicalTrials.gov: NCT01712620.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3653687/

Record 1302

pmcid: PMC3549301

pmid: 22689129

doi: 10.1038/pcan.2012.20

title: A randomized phase II study of pomegranate extract for men with rising PSA following initial therapy for localized prostate cancer.

authors

- Paller CJ

- Ye X
- Wozniak PJ
- Gillespie BK
- Sieber PR
- Greengold RH
- Stockton BR
- Hertzman BL
- Efros MD
- Roper RP
- Liker HR
- Carducci MA

journal: Prostate cancer and prostatic diseases

journalAbbr: Prostate Cancer Prostatic Dis

year: 2013

pubdate: 2013 Mar

volume: 16

issue: 1

pages: 50-5

abstract: BACKGROUND: Pomegranate juice has been associated with PSA doubling time (PSADT) elongation in a single-arm phase II trial. This study assesses biological activity of two doses of pomegranate extract (POMx) in men with recurrent prostate cancer, using changes in PSADT as the primary outcome. METHODS: This randomized, multi-center, double-blind phase II, dose-exploring trial randomized men with a rising PSA and without metastases to receive 1 or 3 g of POMx, stratified by baseline PSADT and Gleason score. Patients (104) were enrolled and treated for up to 18 months. The intent-to-treat (ITT) population was 96% white, with median age 74.5 years and median Gleason score 7. This study was designed to detect a 6-month on-study increase in PSADT from baseline in each arm. RESULTS: Overall, median PSADT in the ITT population lengthened from 11.9 months at baseline to 18.5 months after treatment ($P < 0.001$). PSADT lengthened in the low-dose group from 11.9 to 18.8 months and 12.2 to 17.5 months in the high-dose group, with no significant difference between dose groups ($P = 0.554$). PSADT increases >100% of baseline were observed in 43% of patients. Declining PSA levels were observed in 13 patients (13%). In all, 42% of patients discontinued treatment before meeting the protocol-definition of PSA progression, or 18 months, primarily due to a rising PSA. No significant changes occurred in testosterone. Although no clinically significant toxicities were seen, diarrhea was seen in 1.9% and 13.5% of patients in the 1- and 3-g dose groups, respectively. CONCLUSIONS: POMx treatment was associated with ≥ 6 month increases in PSADT in both treatment arms without adverse effects. The significance of this on-study slowing of PSADT remains unclear, reinforcing the need for placebo-controlled studies in this patient population.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3549301/

Record 1303

pmcid: PMC4063525

pmid: 23809043

doi: 10.1016/j.jpeds.2013.05.033

title: A single histrelin implant is effective for 2 years for treatment of central precocious puberty.

authors

- Lewis KA
- Goldyn AK

- West KW
- Eugster EA

journal: The Journal of pediatrics

journalAbbr: J Pediatr

year: 2013

pubdate: 2013 Oct

volume: 163

issue: 4

pages: 1214-6

abstract: We investigated whether a "yearly" histrelin implant would provide pubertal suppression when left in place for 2 years. Equivalent suppression was observed when comparing 12 and 24 months in 33 children with central precocious puberty. A single implant for 2 years reduces cost and number of implant procedures.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4063525/

Record 1304

pmcid: PMC3654146

pmid: 23625304

doi: 10.1007/s11606-013-2436-y

title: Abstracts of the 36th Annual Meeting of the Society of General Internal Medicine. April 24-27, 2013. Denver, Colorado, USA.

authors

journal: Journal of general internal medicine

journalAbbr: J Gen Intern Med

year: 2013

pubdate: 2013 Jun

volume: 28 Suppl 1

issue: Suppl 1

pages: S1-489

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3654146/

Record 1305

pmcid: PMC3830364

pmid:

doi:

title: ABSTRACTS.

authors

journal: Indian Journal of Endocrinology and Metabolism

journalAbbr: Indian J Endocrinol Metab

year: 2013

pubdate: 2013 Oct

volume: 17

issue: Suppl1

pages: S373-94

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3830364/

Record 1306

pmcid: PMC4173524

pmid: 25885826

doi: 10.4103/0259-1162.118945

title: Anaesthetic challenges and management during pregnancy: Strategies revisited.

authors

- Bajwa SJ
- Bajwa SK

journal: Anesthesia, essays and researches

journalAbbr: Anesth Essays Res

year: 2013

pubdate: 2013 May-Aug

volume: 7

issue: 2

pages: 160-7

abstract: During pregnancy, an obstetrician can encounter various complications and sometimes require surgery or operative intervention for delivery. However, the role of anaesthesiologists during such clinical scenario is grossly under-estimated. Without any close coordination and team work among obstetricians, neonatologists and an anaesthesiologist, morbidity and mortality can increase during these surgical interventions. The clinical scenario can become more challenging if the parturient suffers from any comorbid diseases. The present article reviews some of the common challenging scenarios during pregnancy that an anaesthesiologist frequently encounters during routine practice. Anaesthetic management has been discussed briefly and separately for each trimester and post-partum period. The article also aims at gaining in-depth knowledge of these obstetrical and surgical emergencies so as to ensure close-knit team work among obstetricians, anaesthesiologists, intensivists and a neonatologist.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4173524/

Record 1307

pmcid: PMC4076350

pmid: 23782840

doi: 10.1095/biolreprod.113.109132

title: Androgen receptor (AR) physiological roles in male and female reproductive systems: lessons learned from AR-knockout mice lacking AR in selective cells.

authors

- Chang C
- Lee SO
- Wang RS

- Yeh S
- Chang TM

journal: Biology of reproduction

journalAbbr: Biol Reprod

year: 2013

pubdate: 2013 Jul

volume: 89

issue: 1

pages: 21

abstract: Androgens/androgen receptor (AR) signaling is involved primarily in the development of male-specific phenotypes during embryogenesis, spermatogenesis, sexual behavior, and fertility during adult life. However, this signaling has also been shown to play an important role in development of female reproductive organs and their functions, such as ovarian folliculogenesis, embryonic implantation, and uterine and breast development. The establishment of the testicular feminization (Tfm) mouse model exploiting the X-linked Tfm mutation in mice has been a good in vivo tool for studying the human complete androgen insensitivity syndrome, but this mouse may not be the perfect in vivo model. Mouse models with various cell-specific AR knockout (ARKO) might allow us to study AR roles in individual types of cells in these male and female reproductive systems, although discrepancies are found in results between labs, probably due to using various Cre mice and/or knocking out AR in different AR domains. Nevertheless, no doubt exists that the continuous development of these ARKO mouse models and careful studies will provide information useful for understanding AR roles in reproductive systems of humans and may help us to develop more effective and more specific therapeutic approaches for reproductive system-related diseases.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4076350/

Record 1308

pmcid: PMC3830326

pmid: 24251180

doi: 10.4103/2230-8210.119594

title: Aromatase inhibitors in male sex.

authors

- Singh SK

journal: Indian journal of endocrinology and metabolism

journalAbbr: Indian J Endocrinol Metab

year: 2013

pubdate: 2013 Oct

volume: 17

issue: Suppl 1

pages: S259-61

abstract: Aromatase inhibitors (AI) have been used in males in idiopathic short stature, constitutional delay of puberty, precocious puberty, gynecomastia, oligospermia, hypogonadism related to obesity and ageing. This retrospective study echoed the benefit of AI in Indian males in varied conditions.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3830326/

Record 1309

pmcid: PMC3639656

pmid: 23634285

doi: 10.1002/cam4.48

title: Assessment of diagnosis of inflammatory breast cancer cases at two cancer centers in Egypt and Tunisia.

authors

- Schairer C
- Soliman AS
- Omar S
- Khaled H
- Eissa S
- Ayed FB
- Khalafallah S
- Ayoub WB
- Kantor ED
- Merajver S
- Swain SM
- Gail M
- Brown LM

journal: Cancer medicine

journalAbbr: Cancer Med

year: 2013

pubdate: 2013 Apr

volume: 2

issue: 2

pages: 178-84

abstract: The diagnosis of inflammatory breast cancer (IBC) is largely clinical and therefore inherently somewhat subjective. The objective of this study was to evaluate the diagnosis of IBC at two centers in North Africa where a higher proportion of breast cancer is diagnosed as IBC than in the United States (U.S.). Physicians prospectively enrolled suspected IBC cases at the National Cancer Institute (NCI) - Cairo, Egypt, and the Institut Salah Azaiz (ISA), Tunisia, recorded extent and duration of signs/symptoms of IBC on standardized forms, and took digital photographs of the breast. After second-level review at study hospitals, photographs and clinical information for confirmed IBC cases were reviewed by two U.S. oncologists. We calculated percent agreement between study hospital and U.S. oncologist diagnoses. Among cases confirmed by at least one U.S. oncologist, we calculated median extent and duration of signs and Spearman correlations. At least one U.S. oncologist confirmed the IBC diagnosis for 69% (39/50) of cases with photographs at the NCI-Cairo and 88% (21/24) of cases at the ISA. All confirmed cases had at least one sign of IBC (erythema, edema, peau d'orange) that covered at least one-third of the breast. The median duration of signs ranged from 1 to 3 months; extent and duration of signs were not statistically significantly correlated. From the above-mentioned outcomes, it can be concluded that the diagnosis of a substantial proportion of IBC cases is unambiguous, but a subset is difficult to distinguish from other types of locally advanced breast cancer. Among confirmed cases, the extent of signs was not related to delay in diagnosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3639656/

Record 1310

pmcid: PMC3830363

pmid: 24251217

doi: 10.4103/2230-8210.119628

title: Assessment of self-awareness among rural adolescents: A cross-sectional study.

authors

- Jain RB
- Kumar A
- Khanna P

journal: Indian journal of endocrinology and metabolism

journalAbbr: Indian J Endocrinol Metab

year: 2013

pubdate: 2013 Oct

volume: 17

issue: Suppl 1

pages: S367-72

abstract: CONTEXT: Adolescence is a period of biological, cognitive and social transition of such magnitude and rapidity that it is no surprise to find that it is associated with the onset or exacerbation of a number of health-related problems. It is the level of self-awareness among adolescents, which enables them to see where their thoughts and emotions take them. AIMS: The aim of this study was to assess the extent of awareness regarding adolescent changes/problems among school going adolescents. SETTINGS AND DESIGN: It was a cross-sectional study and was carried out in Block Beri, District, Jhajjar (Haryana). MATERIALS AND METHODS: A sample of 320 adolescent students of 9(th)-12(th) classes (80 from each school) were selected from four randomly chosen large Government senior secondary schools with strength of more than 250 students (two girls and two boys/co-ed senior secondary schools). Data were collected on predesigned, pre-tested and semi-structured schedules by conducting in-depth interviews of selected study adolescents by the investigator. STATISTICAL ANALYSIS USED: Percentages, proportions, Chi-square test, Chi-square test with Yate's correction and t-test. RESULTS: Out of 320, 212 (66.3%) study adolescents were aware of at least one adolescent change(s) whereas, when probed and further asked to enumerate the changes taking place in them, 272/320 (85%) adolescents could narrate at least one such change. Out of those 272, 24 (8.82%) (95% CI 6.0-12.79) adolescents either did not consider these changes as normal or they did not know whether the changes were normal or abnormal. CONCLUSIONS: Adolescents greatly lack correct information related to their bodies' physiological, psychological and sexual changes. There is an urgent need for regular adolescent friendly information, education and communication activities covering different aspects of adolescent knowledge needs/problems.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3830363/

Record 1311

pmcid: PMC3623623

pmid: 23548039

doi: 10.1186/1471-2482-13-9

title: Benign proliferative breast diseases among female patients at a sub-Saharan Africa tertiary hospital: a cross sectional study.

authors

- Okoth C
- Galukande M
- Jombwe J
- Wamala D

journal: BMC surgery

journalAbbr: BMC Surg

year: 2013

pubdate: 2013 Apr 1

volume: 13

issue:

pages: 9

abstract: BACKGROUND: Non-cancerous diseases of the breast have assumed increasing importance because of the public awareness of breast cancer. These benign diseases are a recognized important risk factor for later breast cancer which can develop in either breast. The risk estimate of these benign breast diseases has not been well established in sub Saharan Africa. Women with benign proliferative or atypical breast lesions have a two- fold risk of developing breast cancer in western populations. The purpose of this study therefore was to determine the prevalence of proliferative disease (BPBD) with and without atypia among Ugandan Black women. METHODS: A cross-sectional descriptive study conducted at Mulago Hospital Breast Clinic between January 2012 and June 2012; 208 women aged 12 years and above with palpable breast lumps were screened. Fine needle aspiration biopsies were taken for cytological examination. RESULTS: Of the 208 women with benign breast lumps screened, 195 were recruited in the study. The prevalence of BPBD was 18% (35/195). BPBD with atypia was 5.6% (11/195). The mean age and body mass index (BMI) were 28.4 years and 23.26 kg/m2 respectively. The commonest lesions were fibroadenomas for 57%, (111/195), and fibrocystic change were 21% (40/195). Most BPBD with atypia lesions were in the fibrocystic category. CONCLUSIONS: Benign proliferative breast diseases are common, found mostly among premenopausal women. A significant proportion of BPBD had atypical proliferation. An accurate breast cancer risk estimate study for BPBD is recommended.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3623623/

Record 1312

pmcid: PMC3806122

pmid: 24195000

doi: 10.1155/2013/139276

title: Bilateral primary angiosarcoma of the breast.

authors

- Keshav P
- Hegde SS

journal: Case reports in surgery

journalAbbr: Case Rep Surg

year: 2013

pubdate: 2013

volume: 2013

issue:

pages: 139276

abstract: Primary breast sarcomas are very rare entities, accounting for 0.04% of all malignant neoplasms. Angiosarcoma of breast is infrequent and is an endothelial malignant tumor with bad prognosis because of the frequency of metastasis and recurrence. We present a case of a 30-year-old female who presented with an ulcerated left breast lesion which on further workup revealed to be a primary angiosarcoma of breast with metastasis to right breast.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3806122/

Record 1313

pmcid: PMC3857169

pmid: 24349006

doi: 10.1371/journal.pone.0080616

title: Breast cancer mortality vs. exercise and breast size in runners and walkers.

authors

- Williams PT

journal: PloS one

journalAbbr: PLoS One

year: 2013

pubdate: 2013

volume: 8

issue: 12

pages: e80616

abstract: PURPOSE: Identify predictors of breast cancer mortality in women who exercised below (<7.5 metabolic equivalent hours/week, MET-hours/wk), at (7.5 to 12.5 MET-hours/wk), or above (≥12.5 MET-hours/wk) recommended levels. METHODS: Cox proportional hazard analyses of baseline pre-diagnosis MET-hours/wk vs. breast cancer mortality adjusted for follow-up age, race, baseline menopause, and estrogen and oral contraceptive use in 79,124 women (32,872 walkers, 46,252 runners) from the National Walkers' and Runners' Health Studies. RESULTS: One-hundred eleven women (57 walkers, 54 runners) died from breast cancer during the 11-year follow-up. The decline in mortality in women who exercised ≥7.5 MET-hours/wk was not different for walking and running (P = 0.34), so running and walking energy expenditures were combined. The risk for breast cancer mortality was 41.5% lower for ≥7.5 vs. <7.5 MET-hours/wk (HR: 0.585, 95%CI: 0.382 to 0.924, P = 0.02), which persisted when adjusted for BMI (HR: 0.584, 95%CI: 0.368 to 0.956, P = 0.03). Other than age and menopause, baseline bra cup size was the strongest predictor of breast cancer mortality, i.e., 57.9% risk increase per cup size when adjusted for MET-hours/wk and the other covariates (HR: 1.579, 95%CI: 1.268 to 1.966, P<0.0001), and 70.4% greater when further adjusted for BMI (HR: 1.704, 95%CI: 1.344 to 2.156, P = 10(-5)). Breast cancer mortality was 4.0-fold greater (HR: 3.980, 95%CI: 1.894 to 9.412, P = 0.0001) for C-cup, and 4.7-fold greater (HR: 4.668, 95%CI: 1.963 to 11.980, P = 0.0004) for ≥D-cup vs. A-cup when adjusted for BMI and other covariates. Adjustment for cup size and BMI did not eliminate the association between breast cancer mortality and ≥7.5 MET-hour/wk walked or run (HR: 0.615, 95%CI: 0.389 to 1.004, P = 0.05). CONCLUSION: Breast cancer mortality decreased in association with both meeting the exercise recommendations and smaller breast volume.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3857169/

Record 1314

pmcid: PMC3615620

pmid: 23577249

doi: 10.1155/2013/140704

title: Breast developmental anomalies in dormaa municipality of ghana: prevalence and impact on the life of the individual.

authors

- Agbenorku P
- Otupiri E
- Fugar S

journal: Plastic surgery international

journalAbbr: Plast Surg Int

year: 2013

pubdate: 2013

volume: 2013

issue:

pages: 140704

abstract: Background. Breast developmental anomalies (BDAs) are abnormalities of breast tissue that arise during breast development. Some of the anomalies can have negative impact on the person's life. This study seeks to assess the prevalence of BDA in the Dormaa Municipality in Ghana and its impact on the life of the individual. Materials and Methods. A descriptive study involving 500 female respondents aged between 11 and 25 years from selected schools in the Dormaa Municipality using self-administered questionnaires and interviews. Results. From the study, it was found that the prevalence of BDA in the municipality was 12.8%. The commonest BDA was bilateral hypoplasia which accounted for 31.3% of the BDAs found in the study. Nine (14.1%) complained of the BDA affecting their lives with most being teased in school. Twenty-two (34.4%) girls out of the 64 with BDAs had a family member with a BDA. Conclusion. BDA is a worry; therefore, comprehensive educational programs for health workers and the general public are needed to increase awareness. Also, work should be done to include education on BDA when awareness is being raised about breast cancer and on the importance of breast self-Examination (BSE).

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3615620/

Record 1315

pmcid: PMC3781252

pmid: 23881348

doi: 10.1007/s13244-012-0211-y

title: Breast disease in the pregnant and lactating patient: radiological-pathological correlation.

authors

- Joshi S
- Dialani V
- Marotti J
- Mehta TS
- Slanetz PJ

journal: Insights into imaging

journalAbbr: Insights Imaging

year: 2013

pubdate: 2013 Oct

volume: 4

issue: 5

pages: 527-38

abstract: OBJECTIVE: Substantial physiological changes occur during pregnancy and lactation, making breast evaluation challenging in these patients. This article reviews the imaging challenges of the breast during pregnancy and lactation. The normal imaging appearance, imaging protocols and the imaging features of each commonly encountered benign and malignant entity with pathological correlation and supporting examples is described. An awareness of the imaging features of the breast during these physiological states and of various benign and malignant diseases that occur permits optimal management. CONCLUSIONS: Evaluation of the pregnant and lactating patients who present with a breast problem is challenging. Although ultrasound may

characterise the finding in many cases, mammography and even MRI may have a role in the management of these patients. TEACHING POINTS: • To review physiological changes of the breast during pregnancy and lactation • To review imaging protocols of the breast during pregnancy and lactation • Discuss imaging findings with pathological correlation of benign and malignant diseases in pregnancy and lactation • Discuss pathological correlation of imaging findings in pregnancy and lactation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3781252/

Record 1316

pmcid: PMC3726980

pmid: 23923082

doi:

title: Breast implant-associated ALK-negative anaplastic large cell lymphoma: a case report and discussion of possible pathogenesis.

authors

- George EV
- Pharm J
- Houston C
- Al-Quran S
- Brian G
- Dong H
- Hai W
- Reeves W
- Yang LJ

journal: International journal of clinical and experimental pathology

journalAbbr: Int J Clin Exp Pathol

year: 2013

pubdate: 2013

volume: 6

issue: 8

pages: 1631-42

abstract: Breast implant associated anaplastic large cell lymphoma (BIA-ALCL) is a recently recognized clinical entity, with only 39 well-documented cases reported worldwide, including 3 fatalities. Because of its rarity, the clinical and pathologic features of this malignancy have yet to be fully defined. Moreover, the pathogenesis of ALCL in association with textured silicone gel breast implants is poorly understood. Here we report a case of BIA-ALCL arising in a 67-year-old woman with a mastectomy due to breast cancer followed by implantation of textured silicone gel breast prosthesis. The patient presented with breast enlargement and tenderness 8 years following reconstructive surgery. MRI revealed a fluid collection surrounding the affected breast implant. Pathologic examination confirmed the presence of malignant ALCL T cells that were CD30+, CD8+, CD15+, HLA-DR+, CD25+ ALK- and p53. A diagnosis of indolent BIA-ALCL was made since tumor cells were not found outside of the capsule. Interestingly, an extensive mixed lymphocytic infiltrate and ectopic lymphoid tissue (lymphoid neogenesis) adjacent to the fibrous implant capsule were present. The patient was treated with capsulectomy and implantation of new breast prostheses. Six months later, the patient was found to have BIA-ALCL involvement of an axillary lymph node with cytogenetic evolution of the tumor. To our knowledge, this is the sixth reported case of aggressive BIA-ALCL. Unique features of this case include the association with lymphoid neogenesis and the in vivo cytogenetic progression of the tumor. This case provides insight into the potential role of chronic inflammation and genetic instability in the pathogenesis of BIA-ALCL.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3726980/

Record 1317

pmcid: PMC3812530

pmid: 24198925

doi: 10.4081/pr.2013.e13

title: Central precocious puberty and granulosa cell ovarian tumor in an 8-year old female.

authors

- Calcaterra V
- Nakib G
- Pelizzo G
- Rundo B
- Anna Rispoli G
- Boghen S
- Bonetti F
- Del Monte B
- Gertosio C
- Larizza D

journal: Pediatric reports

journalAbbr: Pediatr Rep

year: 2013

pubdate: 2013

volume: 5

issue: 3

pages: e13

abstract: Ovarian tumors associated with hormonal changes of the peripheral iso-sexual precocious puberty are of common presentation. We describe here a rare case of juvenile granulosa cell tumor in a female with central precocious puberty (CPP). An 8-year old girl with CPP presented with vaginal bleeding four months after the diagnosis and before starting treatment with gonadotropin-releasing hormone (GnRH)-analogs. Suppression of basal follicle-stimulating hormone (FSH) level, elevation of serum estradiol, progesterone and Cancer Antigen-125 were documented. Abdominal ultrasound examination (US) and magnetic resonance imaging showed a pelvic mass affecting the left ovary. A left salpingo-oophorectomy was performed and the mass was totally resected. Juvenile granulosa cell ovarian tumor was diagnosed. One month post surgery, estradiol and progesterone decreased to values of the first evaluation and FSH increased; Cancer Antigen-125 resulted normal while ultrasound pelvic examination showed absence of pelvic masses. In our patient, the tumor had grown very quickly since hormonal data demonstrated a CPP without any evidence of ovarian mass on US only four months before diagnosis. The overstimulation of the FSH or aberrant activation of FSH receptors may have contributed to the development of the mass.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3812530/

Record 1318

pmcid: PMC3658274

pmid: 23632384

doi: 10.1038/msb.2013.20

title: Characterization of drug-induced transcriptional modules: towards drug repositioning and functional

understanding.

authors

- Iskar M
- Zeller G
- Blattmann P
- Campillos M
- Kuhn M
- Kaminska KH
- Runz H
- Gavin AC
- Pepperkok R
- van Noort V
- Bork P

journal: Molecular systems biology

journalAbbr: Mol Syst Biol

year: 2013

pubdate: 2013

volume: 9

issue:

pages: 662

abstract: In pharmacology, it is crucial to understand the complex biological responses that drugs elicit in the human organism and how well they can be inferred from model organisms. We therefore identified a large set of drug-induced transcriptional modules from genome-wide microarray data of drug-treated human cell lines and rat liver, and first characterized their conservation. Over 70% of these modules were common for multiple cell lines and 15% were conserved between the human in vitro and the rat in vivo system. We then illustrate the utility of conserved and cell-type-specific drug-induced modules by predicting and experimentally validating (i) gene functions, e.g., 10 novel regulators of cellular cholesterol homeostasis and (ii) new mechanisms of action for existing drugs, thereby providing a starting point for drug repositioning, e.g., novel cell cycle inhibitors and new modulators of α-adrenergic receptor, peroxisome proliferator-activated receptor and estrogen receptor. Taken together, the identified modules reveal the conservation of transcriptional responses towards drugs across cell types and organisms, and improve our understanding of both the molecular basis of drug action and human biology.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3658274/

Record 1319

pmcid: PMC3706055

pmid: 24872744

doi: 10.1055/s-0033-1343999

title: Cosmetic and reconstructive breast surgery in adolescents: psychological, ethical, and legal considerations.

authors

- Crerand CE
- Magee L

journal: Seminars in plastic surgery

journalAbbr: Semin Plast Surg

year: 2013

pubdate: 2013 Feb

volume: 27

issue: 1

pages: 72-8

abstract: Cosmetic and reconstructive breast surgery has increased in popularity among adolescents in the United States. As more adolescents pursue these procedures, an understanding of the psychological aspects of these surgeries becomes increasingly important for the benefit of both patients and providers. The authors review the psychological aspects of cosmetic and reconstructive surgical breast procedures as they pertain to adolescents, including augmentation mammoplasty, gynecomastia correction, breast reduction, and asymmetry correction. They include a discussion of the medicolegal and ethical implications of these procedures and recommendations for clinical management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3706055/

Record 1320

pmcid: PMC3699085

pmid: 23826050

doi: 10.5489/cuaj.12131

title: Cost-effectiveness of dutasteride-tamsulosin combination therapy for the treatment of symptomatic benign prostatic hyperplasia: A Canadian model based on the CombAT trial.

authors

- Ismaila A
- Walker A
- Sayani A
- Laroche B
- Nickel JC
- Posnett J
- Su Z

journal: Canadian Urological Association journal = Journal de l'Association des urologues du Canada

journalAbbr: Can Urol Assoc J

year: 2013

pubdate: 2013 May-Jun

volume: 7

issue: 5-6

pages: E393-401

abstract: INTRODUCTION: Benign prostatic hyperplasia (BPH) is common in men 50 years old and older. The main treatment options are alpha-blockers (such as tamsulosin), which reduce symptoms, and 5-alpha reductase inhibitors (such as dutasteride), which reduce symptoms and slow disease progression. Clinical studies have demonstrated that dutasteride-tamsulosin combination therapy is more effective than either monotherapy to treat symptomatic BPH. We studied the cost-effectiveness in Canada of the dutasteride (0.5 mg/day) and tamsulosin (0.4 mg/day) combination compared with tamsulosin or dutasteride monotherapy. METHODS: A Markov model was developed which follows a cohort of male BPH patients ≥50 with moderate to severe lower urinary tract symptoms (LUTS). The model estimates costs to the Canadian health care system and outcomes (in terms of quality adjusted life years [QALYs]) at 10 years and over a patient's lifetime. The

dutasteride-tamsulosin combination was compared to each of tamsulosin monotherapy and dutasteride monotherapy. RESULTS: Compared with tamsulosin, the combination was more costly and produced better patient outcomes. Over a lifetime, the incremental cost-effectiveness ratio was CAN$25 437 per QALY gained. At a willingness to pay CAN$50 000 per QALY, the probability of combination therapy being cost-effective was 99.6%. Compared with dutasteride, the combination therapy was the dominant option from year 2, offering improved patient outcomes at lower cost. The probability that combination therapy is more cost-effective than dutasteride was 99.8%. CONCLUSION: Combination therapy offers important clinical benefits for patients with symptomatic BPH, and there is a high probability that it is cost-effective in the Canadian health care system relative to either monotherapy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3699085/

Record 1321

pmcid: PMC3945635

pmid: 24648678

doi: 10.4103/0970-9371.126674

title: Cytodiagnosis of the primary breast lymphoma in a young woman.

authors

- Patne SC
- Kumar M
- Khanna R

journal: Journal of cytology

journalAbbr: J Cytol

year: 2013

pubdate: 2013 Oct

volume: 30

issue: 4

pages: 284-6

abstract: Primary breast lymphoma (PBL) is a rare entity. It represents 0.4% of malignant breast lesions and 2% of extranodal lymphomas. A 23-year-old woman presented with a left breast lump followed by palpable left axillary lymphadenopathy. Fine-needle aspiration cytology (FNAC) and histopathology were diagnostic of non-Hodgkin lymphoma-large cell type. Immunohistochemistry was positive against leukocyte common antigen and CD20. Because PBLs are uncommon malignant lesions and they usually do not have characteristic clinical and imaging findings, FNAC may prove to be a simple, rapid, reliable, and cost-effective procedure for successful diagnosis of the PBL.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3945635/

Record 1322

pmcid: PMC3706056

pmid: 24872732

doi: 10.1055/s-0033-1343989

title: Development of the human breast.

authors

- Javed A
- Lteif A

journal: Seminars in plastic surgery

journalAbbr: Semin Plast Surg

year: 2013

pubdate: 2013 Feb

volume: 27

issue: 1

pages: 5-12

abstract: Mammalia are so named based on the presence of the mammary gland in the breast. The mammary gland is an epidermal appendage, derived from the apocrine glands. The human breast consists of the parenchyma and stroma, originating from ectodermal and mesodermal elements, respectively. Development of the human breast is distinctive for several reasons. The human breast houses the mammary gland that produces and delivers milk through development of an extensive tree-like network of branched ducts. It is also characterized by cellular plasticity, with extensive remodeling in adulthood, a factor that increases its susceptibility to carcinogenesis. Also, breast development occurs in distinct stages via complex epithelial-mesenchymal interactions, orchestrated by signaling pathways under the regulation of systemic hormones. Congenital and acquired disorders of the breast often have a basis in development, making its study essential to understanding breast pathology.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3706056/

Record 1323

pmcid: PMC3776568

pmid: 24058719

doi:

title: Dual pedicle mastopexy technique for reorientation of volume and shape after subglandular and submuscular breast implant removal.

authors

- Gurunluoglu R
- Kubek E
- Arton J

journal: Eplasty

journalAbbr: Eplasty

year: 2013

pubdate: 2013

volume: 13

issue:

pages: e48

abstract: BACKGROUND: The purpose of this article was to report our experience in achieving satisfactory breast shape and volume using dual pedicle mastopexy technique after subglandular and submuscular breast implant removal. METHODS: Breast implant-related problems in 51 breasts included capsular contracture (Baker grade III-IV), 76%; implant rupture/bleed, 41%; breasts undergoing repeat surgery more than once, 22%. The size of the breast implants removed ranged from 240 to 525 cc (average size: 320 ± 65 cc) (saline-filled, 40%; silicone-filled, 60%; subglandular, 40%; submuscular, 60%). Capsulectomy, implant removal, and dual pedicle mastopexy were performed for reconfiguration of breast shape and reorientation of volume. RESULTS: Mean follow-up was 14.5 months. On average, 65.0% of breast implant volume was achieved. There was 1-cup reduction in brassiere size in 21 patients, and the cup size remained the same in 5 patients. Postoperative pain scores were no pain or mild pain in 26 patients who initially in the preoperative evaluation reported having mild pain (11), moderate pain (12), and severe pain (3). Overall patient satisfaction scores were 3 (neutral) in 1

patient, 4 (satisfied) in 12 patients, and 5 (very satisfied) in 13 patients. CONCLUSION: The dual pedicle mastopexytechnique provided a reliable way of reorienting breast volume and configuring breast shape in patients who opted to have implants removed without replacement. The results demonstrated that a pleasing outcome could be obtained using the described technique with additional benefits of elimination of breast tenderness and discomfort secondary to implant removal and/or capsulectomy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3776568/

Record 1324

pmcid: PMC3751361

pmid: 23941256

doi: 10.1186/1471-2431-13-120

title: Efavirenz-induced gynecomastia in a prepubertal girl with human immunodeficiency virus infection: a case report.

authors

- van Ramshorst MS
- Kekana M
- Struthers HE
- McIntyre JA
- Peters RP

journal: BMC pediatrics

journalAbbr: BMC Pediatr

year: 2013

pubdate: 2013 Aug 13

volume: 13

issue:

pages: 120

abstract: BACKGROUND: Prepubertal gynecomastia is a rare condition and most frequently classified as idiopathic. In HIV-infected adults gynecomastia is a recognised but infrequent side-effect of antiretroviral treatment (ART) and mostly attributed to efavirenz use. Gynecomastia should be distinguished from pseudogynecomastia as part of the lipodystrophy syndrome caused by Nucleoside Reverse Transcriptase Inhibitors (NRTIs) to avoid incorrect substitution of drugs. In the medical literature only five cases of prepubertal gynecomastia in children taking ART are described and underlying pathogenesis was unknown. The occurrence of adverse effects of ART may interfere with therapy adherence and long-term prognosis and for that reason requires attention. We report the first case of prepubertal gynecomastia in a young girl attributed to efavirenz use. CASE PRESENTATION: A seven-year-old African girl presented with true gynecomastia four months after initiation on ART (abacavir, lamivudine, efavirenz). History, physical examination and laboratory tests excluded known causes of gynecomastia and efavirenz was considered as the most likely cause. Six weeks after withdrawal of efavirenz the breast enlargement had completely resolved. CONCLUSIONS: Efavirenz-induced gynecomastia may occur in children as well as in adults. With the increasing access to ART, the possibility of efavirenz-exposure and the potential occurrence of its associated side-effects may be high. In resource-poor settings, empirical change from efavirenz to nevirapine may be considered, providing no other known or alarming cause is identified, as efavirenz-induced gynecomastia can resolve quickly after withdrawal of the drug. Timely recognition of gynecomastia as a side-effect of efavirenz is important in order to intervene while the condition may still be reversible, to sustain adherence to ART and to maintain the sociopsychological health of the child.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3751361/

Record 1325

pmcid: PMC3767933

pmid: 24011889

doi: 10.1016/j.ecl.2013.05.008

title: Effects of alcohol on the endocrine system.

authors

- Rachdaoui N
- Sarkar DK

journal: Endocrinology and metabolism clinics of North America

journalAbbr: Endocrinol Metab Clin North Am

year: 2013

pubdate: 2013 Sep

volume: 42

issue: 3

pages: 593-615

abstract: Chronic consumption of a large amount of alcohol disrupts the communication between nervous, endocrine, and immune system and causes hormonal disturbances that lead to profound and serious consequences at physiologic and behavioral levels. These alcohol-induced hormonal dysregulations affect the entire body and can result in various disorders such as stress abnormalities, reproductive deficits, body growth defect, thyroid problems, immune dysfunction, cancers, bone disease, and psychological and behavioral disorders. This review summarizes the findings from human and animal studies that provide consistent evidence on the various effects of alcohol abuse on the endocrine system.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3767933/

Record 1326

pmcid: PMC3839433

pmid: 24421423

doi: 10.1310/hpj4801-48

title: Elvitegravir/Cobicistat/Emtricitabine/Tenofovir disoproxil fumarate tablets.

authors

- Cada DJ
- Torres S
- Levien TL
- Baker DE

journal: Hospital pharmacy

journalAbbr: Hosp Pharm

year: 2013

pubdate: 2013 Jan

volume: 48

issue: 1

pages: 48-56

abstract: Each month, subscribers to The Formulary Monograph Service receive 5 to 6 well-documented monographs on drugs that are newly released or are in late phase 3 trials. The monographs are targeted to

Pharmacy & Therapeutics Committees. Subscribers also receive monthly 1-page summary monographs on agents that are useful for agendas and pharmacy/nursing in-services. A comprehensive target drug utilization evaluation/medication use evaluation (DUE/MUE) is also provided each month. With a subscription, the monographs are sent in print and are also available on-line. Monographs can be customized to meet the needs of a facility. Subscribers to The Formulary Monograph Service also receive access to a pharmacy bulletin board, The Formulary Information Exchange (The F.I.X.). All topics pertinent to clinical and hospital pharmacy are discussed on The F.I.X. A drug class review is now published monthly with The Formulary Monograph Service. Through the cooperation of The Formulary, Hospital Pharmacy publishes selected reviews in this column. For more information about The Formulary Monograph Service or The F.I.X., call The Formulary at 800-322-4349. The January 2013 monograph topics are onperampanel, omacetaxine mepesuccinate, ocriplasmin, sodium picosulfate/magnesium oxide/anhydrous citric acid, and lomitapide. The DUE/MUE is on perampanel.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3839433/

Record 1327

pmcid: PMC3748390

pmid: 23953804

doi: 10.1016/j.semnephrol.2013.04.007

title: Endocrine and hypertensive disorders of potassium regulation: primary aldosteronism.

authors

- Weiner ID

journal: Seminars in nephrology

journalAbbr: Semin Nephrol

year: 2013

pubdate: 2013 May

volume: 33

issue: 3

pages: 265-76

abstract: The identification of primary aldosteronism as a common cause of resistant hypertension is a significant advance in our ability to care for patients with hypertension. Primary aldosteronism is common, and when unrecognized is associated with an increased incidence of adverse cardiovascular outcomes. Identification of primary aldosteronism is based on use of the plasma aldosterone level, plasma renin activity, and the aldosterone:renin ratio. Differentiation between unilateral and bilateral autonomous adrenal aldosterone production then guides further therapy, with use of mineralocorticoid-receptor blockers for patients with bilateral autonomous adrenal aldosterone production and laparoscopic adrenalectomy for patients with unilateral autonomous aldosterone production. In this review, we discuss in detail the pathogenesis of primary aldosteronism-induced hypertension and potassium disorders, the evaluation of the patient with suspected primary aldosteronism, and the management of primary aldosteronism, both through medications and surgery.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3748390/

Record 1328

pmcid: PMC3682794

pmid: 23417729

doi: 10.1007/s10911-013-9275-7

title: Endocrine disruptors and the breast: early life effects and later life disease.

authors

- Macon MB

- Fenton SE

  journal: Journal of mammary gland biology and neoplasia

  journalAbbr: J Mammary Gland Biol Neoplasia

  year: 2013

  pubdate: 2013 Mar

  volume: 18

  issue: 1

  pages: 43-61

  abstract: Breast cancer risk has both heritable and environment/lifestyle components. The heritable component is a small contribution (5-27 %), leaving the majority of risk to environment (e.g., applied chemicals, food residues, occupational hazards, pharmaceuticals, stress) and lifestyle (e.g., physical activity, cosmetics, water source, alcohol, smoking). However, these factors are not well-defined, primarily due to the enormous number of factors to be considered. In both humans and rodent models, environmental factors that act as endocrine disrupting compounds (EDCs) have been shown to disrupt normal mammary development and lead to adverse lifelong consequences, especially when exposures occur during early life. EDCs can act directly or indirectly on mammary tissue to increase sensitivity to chemical carcinogens or enhance development of hyperplasia, beaded ducts, or tumors. Protective effects have also been reported. The mechanisms for these changes are not well understood. Environmental agents may also act as carcinogens in adult rodent models, directly causing or promoting tumor development, typically in more than one organ. Many of the environmental agents that act as EDCs and are known to affect the breast are discussed. Understanding the mechanism(s) of action for these compounds will be critical to prevent their effects on the breast in the future.

  url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3682794/

Record 1329

pmcid: PMC3702723

pmid: 23837054

doi: 10.3892/etm.2013.1032

title: Endoscopic subcutaneous mastectomy: A novel and effective treatment for gynecomastia.

authors

- Cao H
- Yang ZX
- Sun YH
- Wu HR
- Jiang GQ

  journal: Experimental and therapeutic medicine

  journalAbbr: Exp Ther Med

  year: 2013

  pubdate: 2013 Jun

  volume: 5

  issue: 6

  pages: 1683-1686

  abstract: The aim of this study was to evaluate the procedure for and efficacy of endoscopic subcutaneous mastectomy for gynecomastia. Endoscopic subcutaneous mastectomy was performed on 100 benign, palpable breast enlargements in 58 male patients who were followed-up for 15-63 months. The surgery was conducted

with the insufflation of CO 2 subdermally. No cases were converted to open surgery. The unilateral surgery time was 70-90 min. The mean volume of the resected tissue was 200 ml. All procedures were completed successfully, with satisfactory clinical effects and ideal esthetic results postoperatively. There were three cases (3%) of papillary epidermal partial necrosis; following removal of the dressing during the hospital stay, normal nipple sensation returned. Endoscopic subcutaneous mastectomy had good clinical effects and ideal cosmetic results and is an appropriate approach for gynecomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3702723/

Record 1330

pmcid: PMC3712386

pmid: 23869312

doi: 10.4103/2230-8210.111669

title: Feminizing adrenal tumors: Our experience about three cases.

authors

- Chentli F
- Bekkaye I
- Yahiaoui S
- Souidi S
- Fedala NS
- Azzoug S

journal: Indian journal of endocrinology and metabolism

journalAbbr: Indian J Endocrinol Metab

year: 2013

pubdate: 2013 May

volume: 17

issue: 3

pages: 509-13

abstract: Feminizing adrenal tumors (FATs) are very rare as they account for less than 2% of all the adrenal neoplasms. Their prognosis is deemed to be very poor. We aimed to present a mono centre (adult and pediatric) experience over a long period of time (January 1980 to Jun 2012). During the study period, we observed only three cases in men aged 22 (2 cases) and 45 (1 case). They all consulted for a painful gynecomastia, decreased libido and impotency. Estradiol was high in two cases at presentation, and after a relapsing tumor in the third one. All had big adrenal tumors (5.9, 6, and 17 cm), and a mixed secretion composed by high estradiol and cortisol. The pathological study argued for malignancy in two cases. But, only one had diffuse metastasis and died 4 years after diagnosis; the others diagnosed one and three years ago are still alive without any metastasis or relapsing.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3712386/

Record 1331

pmcid: PMC3665869

pmid: 23730601

doi: 10.5999/aps.2013.40.3.244

title: Giant lipoma of the breast.

authors

- Ramírez-Montaño L
- Vargas-Tellez E
- Dajer-Fadel WL
- Espinosa Maceda S

journal: Archives of plastic surgery

journalAbbr: Arch Plast Surg

year: 2013

pubdate: 2013 May

volume: 40

issue: 3

pages: 244-6

abstract: Lipomas are benign mesenchymal tumors that develop in areas of abundant adipose tissue. Due to the fatty composition of the breast, difficulties in diagnosis, treatment, and reconstruction are often encountered. We report a case of a 55-year-old female with a giant tumor of the right breast that comprised most of its mass, causing breast asymmetry. A thorough preoperative evaluation, followed by an uneventful difficult surgical resection and reconstruction, resulted in diagnosis of a benign lipoma. The case prompted this report because of its challenging size, location, diagnosis, and reconstructive solution.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3665869/

Record 1332

pmcid: PMC3706045

pmid: 24872741

doi: 10.1055/s-0033-1347166

title: Gynecomastia in adolescent males.

authors

- Lemaine V
- Cayci C
- Simmons PS
- Petty P

journal: Seminars in plastic surgery

journalAbbr: Semin Plast Surg

year: 2013

pubdate: 2013 Feb

volume: 27

issue: 1

pages: 56-61

abstract: Gynecomastia is defined as an enlargement of the male breast. It is often benign, and can be the source of significant embarrassment and psychological distress. A general medical history and careful physical examination are essential to distinguish normal developmental variants from pathological causes. Treatment is geared toward the specific etiology when identified. In the majority of cases of pubertal gynecomastia, observation and reassurance are the mainstays of therapy as the condition usually resolves naturally. Pharmacological treatment and surgery are recommended only in selected cases.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3706045/

Record 1333

pmcid: PMC3795580

pmid: 24171143

doi: 10.4088/PCC.12l01454

title: Hirsutism in a female adolescent induced by long-acting injectable risperidone: a case report.

authors

- Patel A
- Malek N
- Haq F
- Turnbow L
- Raza S

journal: The primary care companion for CNS disorders

journalAbbr: Prim Care Companion CNS Disord

year: 2013

pubdate: 2013

volume: 15

issue: 3

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3795580/

Record 1334

pmcid: PMC6485821

pmid: 23543528

doi: 10.1002/14651858.CD003990.pub4

title: Hormonal versus non-hormonal contraceptives in women with diabetes mellitus type 1 and 2.

authors

- Visser J
- Snel M
- Van Vliet HA

journal: The Cochrane database of systematic reviews

journalAbbr: Cochrane Database Syst Rev

year: 2013

pubdate: 2013 Mar 28

volume: 2013

issue: 3

pages: CD003990

abstract: BACKGROUND: Adequate contraceptive advice is important in both women with diabetes mellitus type 1 and type 2 to reduce the risk of maternal and infant morbidity and mortality in unplanned pregnancies. A wide variety of contraceptives are available for these women. However, hormonal contraceptives might influence carbohydrate and lipid metabolism and increase micro- and macrovascular complications, so caution

in selecting a contraceptive method is required. OBJECTIVES: To investigate whether progestogen-only, combined estrogen and progestogen or non-hormonal contraceptives differ in terms of effectiveness in preventing pregnancy, in their side effects on carbohydrate and lipid metabolism, and in long-term complications such as micro- and macrovascular disease when used in women with diabetes mellitus. SEARCH METHODS: The search was performed in CENTRAL, MEDLINE, EMBASE, POPLINE, CINAHL, WorldCat, ECO, ArticleFirst, the Science Citation Index, the British Library Inside, and reference lists of relevant articles. The last search was performed in January 2013. In addition, experts in the field and pharmaceutical companies marketing contraceptives were contacted to identify published, unpublished or ongoing studies. SELECTION CRITERIA: Randomised and quasi-randomised controlled trials that studied women with diabetes mellitus comparing: 1. hormonal versus non-hormonal contraceptives; 2. progestogen-only versus estrogen and progestogen contraceptives; 3. contraceptives containing < 50 µg estrogen versus contraceptives containing ≥ 50 µg estrogen; and 4. contraceptives containing first-, second- and third-generation progestogens, drospirenone and cyproterone acetate. The principal outcomes were contraceptive effectiveness, diabetes control, lipid metabolism and micro- and macrovascular complications. DATA COLLECTION AND ANALYSIS: Two investigators evaluated the titles and abstracts identified from the literature search. Quality assessment was performed independently with discrepancies resolved by discussion or consulting a third review author. Because the trials differed in studied contraceptives, participant characteristics and methodological quality, we could not combine the data in a meta-analysis. The trials were therefore examined on an individual basis and narrative summaries were provided. MAIN RESULTS: Four randomised controlled trials were included. No unintended pregnancies were reported during the study periods. Only one trial was of good methodological quality. It compared the influence of a levonorgestrel-releasing intrauterine device (IUD) versus a copper IUD on carbohydrate metabolism in women with type 1 diabetes mellitus. No significant difference was found between the two groups. The other three trials were of limited methodological quality. Two compared progestogen-only pills with different estrogen and progestogen combinations, and one also included the levonorgestrel-releasing IUD and copper IUD. The trials reported that blood glucose levels remained stable during treatment with most regimens. Only high-dose combined oral contraceptives and 30 µg ethinylestradiol + 75 µg gestodene were identified as slightly impairing glucose homeostasis. The three studies found conflicting results regarding lipid metabolism. Some combined oral contraceptives appeared to have a minor adverse effect while others appeared to slightly improve lipid metabolism. The copper IUD and progestogen-only oral contraceptives also slightly improved lipid metabolism and no influence was seen while using the levonorgestel-releasing IUD. Only one study reported on micro- and macrovascular complications. It observed no signs or symptoms of thromboembolic incidents or visual disturbances, however study duration was short. Only minor adverse effects were reported in two studies. AUTHORS' CONCLUSIONS: The four included randomised controlled trials in this systematic review provided insufficient evidence to assess whether progestogen-only and combined contraceptives differ from non-hormonal contraceptives in diabetes control, lipid metabolism and complications. Three of the four studies were of limited methodological quality, sponsored by pharmaceutical companies and described surrogate outcomes. Ideally, an adequately reported, high-quality randomised controlled trial analysing both intermediate outcomes (that is glucose and lipid metabolism) and true clinical endpoints (micro- and macrovascular disease) in users of combined, progestogen-only and non-hormonal contraceptives should be conducted. However, due to the low incidence of micro- and macrovascular disease and accordingly the large sample size and long follow-up period needed to observe differences in risk, a randomised controlled trial might not be the ideal design.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6485821/

Record 1335

pmcid: PMC3706054

pmid: 24872740

doi: 10.1055/s-0033-1347167

title: Hyperplastic breast anomalies in the female adolescent breast.

authors

- Wolfswinkel EM
- Lemaine V

- Weathers WM
- Chike-Obi CJ
- Xue AS
- Heller L

journal: Seminars in plastic surgery

journalAbbr: Semin Plast Surg

year: 2013

pubdate: 2013 Feb

volume: 27

issue: 1

pages: 49-55

abstract: Macromastia in adolescents is multifactorial and usually idiopathic, associated with obesity or hormonal imbalances. Less commonly, it can result from virginal or juvenile breast hypertrophy, a rare condition of unknown etiology, where an alarmingly rapid breast enlargement occurs during puberty. Breast hypertrophy in the adolescent population can have significant long-term medical and psychological impacts. Although symptoms can be severe, many plastic surgeons, pediatricians, and parents are often reluctant to surgically treat adolescent macromastia. However, reduction mammoplasty is a safe and effective treatment and may be the only way to alleviate the increased social, psychological, and physical strain caused by macromastia in adolescents.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3706054/

Record 1336

pmcid: PMC3891095

pmid: 24431950

doi: 10.1177/229255031302100209

title: Implant-associated anaplastic large cell lymphoma of the breast: Insight into a poorly understood disease.

authors

- Weathers WM
- Wolfswinkel EM
- Hatef DA
- Lee EI
- Hollier LH
- Brown RH

journal: The Canadian journal of plastic surgery = Journal canadien de chirurgie plastique

journalAbbr: Can J Plast Surg

year: 2013

pubdate: 2013 Summer

volume: 21

issue: 2

pages: 95-8

abstract: Implant-associated anaplastic large cell lymphoma (ALCL) is the subject of much debate in the field of plastic surgery. Only a few published cases have been reported and the rarity of the disease may make proving causality exceedingly difficult. Despite this, it is of utmost importance that full attention be devoted to this subject

to ensure the safety and well-being of patients. The authors report one new case of implant-associated ALCL that recently presented to their institution. Implant-associated ALCL is a poorly understood disease. It should likely be considered its own clinical entity and categorized into two subtypes: one presenting as a seroma and the other as a distinct mass or masses. When reported, only textured implants have been associated with ALCL. The United States Food and Drug Administration and American Society of Plastic Surgeons have initiated a registry and have collected critical data to gain further understanding of this disease.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3891095/

Record 1337

pmcid: PMC3603731

pmid: 23543972

doi: 10.1080/08998280.2013.11928940

title: Inflammatory breast carcinoma.

authors

- Ha KY
- Glass SB
- Laurie L

journal: Proceedings (Baylor University. Medical Center)

journalAbbr: Proc (Bayl Univ Med Cent)

year: 2013

pubdate: 2013 Apr

volume: 26

issue: 2

pages: 149-51

abstract: Inflammatory breast carcinoma is a rare form of invasive breast cancer often characterized by erythema, warmth, and a classic "peau de orange" or "orange peel" appearance of the affected breast. The average age of onset is within the fourth and fifth decades. Lesions are usually detected and evaluated with mammography, sonography, and recently, breast magnetic resonance imaging. We present the case of a 49-year-old woman with inflammatory breast carcinoma in her left breast and describe the imaging appearance of this aggressive lesion on the modalities listed above. Because this lesion may be misdiagnosed as infection (i.e., mastitis) or as the sequelae of a dermatologic disorder, proper characterization of inflammatory breast carcinoma is of the utmost clinical and radiologic importance.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3603731/

Record 1338

pmcid: PMC4899549

pmid: 27330647

doi: 10.2484/rcr.v8i4.855

title: Initial presentation and recurrence of metastatic rhabdomyosarcoma as breast mass.

authors

- Nguyen HV
- Aminololama-Shakeri S
- Zhang Y

journal: Radiology case reports

journalAbbr: Radiol Case Rep

year: 2013

pubdate: 2013

volume: 8

issue: 4

pages: 855

abstract: Rhabdomyosarcoma rarely metastasizes to the breast. We report a case of a pediatric patient who initially presented with a right breast mass and pancytopenia, which was subsequently diagnosed as alveolar rhabdomyosarcoma. Despite initial favorable response to chemotherapy, a new metastatic focus was found in the contralateral breast 10 months later.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4899549/

Record 1339

pmcid: PMC3189440

pmid: 21726261

doi: 10.1111/j.1467-8519.2011.01892.x

title: Is payment a benefit?

authors

- Wertheimer A

journal: Bioethics

journalAbbr: Bioethics

year: 2013

pubdate: 2013 Feb

volume: 27

issue: 2

pages: 105-16

abstract: What I call 'the standard view' claims that IRBs should not regard financial payment as a benefit to subjects for the purpose of risk/benefit assessment. Although the standard view is universally accepted, there is little defense of that view in the canonical documents of research ethics or the scholarly literature. This paper claims that insofar as IRBs should be concerned with the interests and autonomy of research subjects, they should reject the standard view and adopt 'the incorporation view.' The incorporation view is more consistent with the underlying soft-paternalist justification for risk-benefit assessment and demonstrates respect for the autonomy of prospective subjects. Adoption of the standard view precludes protocols that advance the interests of subjects, investigators, and society. After considering several objections to the argument, I consider several arguments for the standard view that do not appeal to the interests and autonomy of research subjects.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3189440/

Record 1340

pmcid: PMC3790613

pmid: 23940128

doi: 10.1210/jc.2013-2411

title: Isolated prolactin deficiency associated with serum autoantibodies against prolactin-secreting cells.

authors

- Iwama S
- Welt CK
- Romero CJ
- Radovick S
- Caturegli P

journal: The Journal of clinical endocrinology and metabolism

journalAbbr: J Clin Endocrinol Metab

year: 2013

pubdate: 2013 Oct

volume: 98

issue: 10

pages: 3920-5

abstract: CONTEXT: Isolated prolactin (PRL) deficiency is a rare entity of unknown etiology manifesting as failure of puerperal lactogenesis. OBJECTIVE: The aim of the study was to determine the cause of isolated PRL deficiency in an affected woman. DESIGN AND SETTING: We examined genetic and autoimmune causes of isolated PRL deficiency at academic medical centers. PATIENT: The patient was a 39-year-old woman with puerperal alactogenesis after two deliveries and undetectable PRL. The other pituitary axes, serum calcium levels, and cranial magnetic resonance imaging were normal. INTERVENTION: Recombinant human PRL (r-hPRL) was administered to the patient. MAIN OUTCOME MEASURES: We measured the sequencing of candidate genes and immunofluorescence analysis of autoantibodies directed against pituitary endocrine cells. RESULTS: There were no rare sequence variants in the genes encoding for PRL, putative PRL-releasing peptide, putative PRL-releasing peptide receptor, or in other genes important for lactotroph lineage development (POU1F1, PROP1, LHX3, LHX4, HESX1, OTX2, and LSD1). The patient serum, on the contrary, contained autoantibodies that specifically recognized a subset of PRL-secreting cells but not PRL itself or any other pituitary cells or hormones. The mother was able to lactate fully after 17 days of treatment with r-hPRL 60 μg/kg every 12 hours, but alactogenesis resumed after treatment was completed. CONCLUSIONS: These studies report a new autoimmune etiology for women with isolated PRL deficiency and puerperal alactogenesis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3790613/

Record 1341

pmcid: PMC3862043

pmid: 24771897

doi: 10.1055/s-0033-1350881

title: Living with Kallmann Syndrome - Analysis of Subjective Experience Reports from Women.

authors

- Hofmann J
- Watzlawik M
- Richter-Appelt H

journal: Geburtshilfe und Frauenheilkunde

journalAbbr: Geburtshilfe Frauenheilkd

year: 2013

pubdate: 2013 Nov

volume: 73

issue: 11

pages: 1112-1120

abstract: Five women were questioned on their experiences with Kallmann Syndrome (KS) in thematically focused, open interviews. This investigation complements the study of the experiences of men with KS 1. The results show that the consequences and pressures of KS extend beyond the somato-medical field and that those affected are also burdened by mental and psycho-social impacts. The pressures experienced by those affected include a distorted body image resulting from the lack of physical development, which in turn leads to difficulties in developing a healthy feeling of self-worth. Furthermore, particular attention should be paid to the influence of hormone therapy on mood and libido during medical treatment. Some of the affected women reported experiencing depressive moods and low libido, and pressures in their relationship associated with this. The affected women wanted KS to be viewed as a whole in order to achieve better handling. In particular, attentive handling of issues relating to fertility was important to them. Additional relevant support included offers of confidential discussions, and offers of psychotherapy and, where required, sex education or sex therapy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3862043/

Record 1342

pmcid: PMC4150593

pmid: 25688049

doi: 10.1016/j.ebcr.2012.10.006

title: Low-dose lacosamide-induced atrial fibrillation: Case analysis with literature review.

authors

- Kaufman KR
- Velez AE
- Wong S
- Mani R

journal: Epilepsy & behavior case reports

journalAbbr: Epilepsy Behav Case Rep

year: 2013

pubdate: 2013

volume: 1

issue:

pages: 22-5

abstract: Lacosamide (LCM) is a novel antiepileptic drug (AED) approved by the FDA for adjunctive treatment of partial epilepsy with and without secondary generalization. Lacosamide dose-dependent dysrhythmias (PR-interval prolongation, AV block, and atrial fibrillation/flutter) have been reported. This case represents the first instance of LCM-induced atrial fibrillation following a low loading dose (200 mg). Risk factors for atrial fibrillation are addressed and discussed in the context of this case. Full cardiac history is recommended prior to patients being initiated on LCM. Cardiac monitoring may be required for at-risk patients on LCM. Clinicians need to be cognizant of this potential adverse effect.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4150593/

Record 1343

pmcid: PMC3603730

pmid: 23543971

doi: 10.1080/08998280.2013.11928939

title: Lymphoma in the breast.

authors

- Ha KY
- Wang JC
- Gill JI

journal: Proceedings (Baylor University. Medical Center)

journalAbbr: Proc (Bayl Univ Med Cent)

year: 2013

pubdate: 2013 Apr

volume: 26

issue: 2

pages: 146-8

abstract: Lymphoma is a rare neoplasm in the breast. In this location, it may be primary or secondary, depending on whether there is lymphoma elsewhere in the body. The most common presentation of breast lymphoma is a painless palpable mass, indistinguishable from that of breast carcinoma, although the treatment regimens for these two neoplasms differ vastly. Knowledge of the varied mammographic and sonographic presentations of breast lymphoma should prompt more frequent recognition of this unusual malignant entity. Proper diagnosis of this neoplasm is of the utmost importance to guide appropriate treatment planning and prevent unnecessary and potentially harmful surgery. We describe secondary breast lymphoma in a woman who had been diagnosed and treated for non-Hodgkin's lymphoma several years earlier.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3603730/

Record 1344

pmcid: PMC3570947

pmid: 23424695

doi: 10.1155/2013/386725

title: Massive ascites as the only sign of ovarian juvenile granulosa cell tumor in an adolescent: a case report and a review of the literature.

authors

- Ashnagar A
- Alavi S
- Nilipour Y
- Azma R
- Falahati F

journal: Case reports in oncological medicine

journalAbbr: Case Rep Oncol Med

year: 2013

pubdate: 2013

volume: 2013

issue:

pages: 386725

abstract: Ovarian neoplasms are relatively rare in childhood and adolescence; only 5% to 8% of the cases are of sex cord stromal origin. Granulosa cell tumors are a group of estrogen producing sex cord stromal tumors of the ovary. They occur in 95% of the cases in adults, and only about 5% of the cases, which differ in histologic

characteristics, are of juvenile type. A 13-year-old girl is reported who presented with massive abdominal distention and ascites. An abdominopelvic computed tomography scan showed a predominantly cystic mass lesion with septations arising from the left ovary. All tumor markers were normal, but serum inhibin level was increased. The patient underwent mass resection with salpingoophorectomy. Histopathology was compatible with the juvenile granulosa cell tumor. Interestingly, menarche was started in the patient soon after the surgery. To the best of our knowledge, massive ascites as the only clinical manifestation in the juvenile granulosa cell tumor has not reported as yet.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3570947/

Record 1345

pmcid: PMC3654311

pmid: 23720649

doi: 10.3389/fneur.2013.00053

title: Mechanisms mediating spinal and bulbar muscular atrophy: investigations into polyglutamine-expanded androgen receptor function and dysfunction.

authors

- Beitel LK
- Alvarado C
- Mokhtar S
- Paliouras M
- Trifiro M

journal: Frontiers in neurology

journalAbbr: Front Neurol

year: 2013

pubdate: 2013

volume: 4

issue:

pages: 53

abstract: Spinal and bulbar muscular atrophy (SBMA, Kennedy's disease), a late-onset neuromuscular disorder, is caused by expansion of the polymorphic polyglutamine tract in the androgen receptor (AR). The AR is a ligand-activated transcription factor, but plays roles in other cellular pathways. In SBMA, selective motor neuron degeneration occurs in the brainstem and spinal cord, thus the causes of neuronal dysfunction have been studied. However, pathogenic pathways in muscles may also be involved. Cultured cells, fly and mouse models are used to study the molecular mechanisms leading to SBMA. Both the structure of the polyglutamine-expanded AR (polyQ AR) and its interactions with other proteins are altered relative to the normal AR. The ligand-dependent translocation of the polyQ AR to the nucleus appears to be critical, as are interdomain interactions. The polyQ AR, or fragments thereof, can form nuclear inclusions, but their pathogenic or protective nature is unclear. Other data suggests soluble polyQ AR oligomers can be harmful. Post-translational modifications such as phosphorylation, acetylation, and ubiquitination influence AR function and modulate the deleterious effects of the polyQ AR. Transcriptional dysregulation is highly likely to be a factor in SBMA; deregulation of non-genomic AR signaling may also be involved. Studies on polyQ AR-protein degradation suggest inhibition of the ubiquitin proteasome system and changes to autophagic pathways may be relevant. Mitochondrial function and axonal transport may also be affected by the polyQ AR. Androgens, acting through the AR, can be neurotrophic and are important in muscle development; hence both loss of normal AR functions and gain of novel harmful functions by the polyQ AR can contribute to neurodegeneration and muscular atrophy. Thus investigations into polyQ AR function have shown that multiple complex mechanisms lead to the initiation and progression of SBMA.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3654311/

Record 1346

pmcid: PMC4533934

pmid: 23220209

doi: 10.1136/archdischild-2012-302633

title: Metabolic abnormalities and body composition of HIV-infected children on Lopinavir or Nevirapine-based antiretroviral therapy.

authors

- Arpadi S
- Shiau S
- Strehlau R
- Martens L
- Patel F
- Coovadia A
- Abrams EJ
- Kuhn L

journal: Archives of disease in childhood

journalAbbr: Arch Dis Child

year: 2013

pubdate: 2013 Apr

volume: 98

issue: 4

pages: 258-64

abstract: BACKGROUND: Few studies have assessed metabolic and body composition alterations in perinatally HIV-infected African children on antiretroviral therapy (ART). We compared metabolic profiles and regional fat of children on ritonavir-boosted lopinavir (lopinavir/ritonavir), lamivudine and stavudine to those switched to nevirapine, lamivudine and stavudine. METHODS: This study evaluated metabolic and body composition outcomes in 156 HIV-infected children completing a randomised trial that assessed the continued use of lopinavir/ritonavir-based ART or switch to nevirapine-based ART in Johannesburg, South Africa (2005-2010). Fasting total cholesterol (TC), high-density lipoprotein (HDL), low-density lipoprotein (LDL), triglycerides total and regional body fat (BF) were measured. A clinical assessment for lipodystrophy (LD) was conducted. RESULTS: 156 children (mean age 5.1±0.8 years, mean duration of treatment 4.2±0.7 years, mean time since randomisation 3.4±0.7 years) were enrolled. 85 were randomised to the lopinavir/ritonavir group and 71 to the nevirapine group. The lopinavir/ritonavir group had lower mean HDL (1.3±0.4 vs 1.5±0.4 mmol/l, p<0.001) and higher mean TC (4.4±1.0 vs 4.1±0.8 mmol/l, p=0.097), LDL (2.6±0.9 vs 2.3±0.7 mmol/l, p=0.018) and triglycerides (1.1±0.4 vs 0.8±0.3 mmol/l, p<0.001). The lopinavir/ritonavir group had more total BF by mean skinfold sum (43±11.1 vs 39±10.1 mm, p=0.031) and BF% by bioelectrical impedance analysis (17.0±7.0 vs 14.1±8.0%, p=0.022). Thirteen (8.4%) met criteria for LD. CONCLUSIONS: Unfavourable alterations in lipid profile and triglycerides, and differences in fat are detectable in young HIV-infected South African children receiving lopinavir/ritonavir-based regimens versus those switched to nevirapine-based regimens. Interventions to mitigate these alterations are warranted to reduce long-term cardiovascular disease risk.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4533934/

Record 1347

pmcid: PMC4027087

pmid: 24904873

doi: 10.6065/apem.2013.18.4.173

title: Multicenter clinical trial of leuprolide acetate depot (Luphere depot 3.75 mg) for efficacy and safety in girls with central precocious puberty.

authors

- Kim YJ
- Lee HS
- Lee YJ
- Lim JS
- Kim SY
- Kim EY
- Jin DK
- Hwang IT
- Hwang JS

journal: Annals of pediatric endocrinology & metabolism

journalAbbr: Ann Pediatr Endocrinol Metab

year: 2013

pubdate: 2013 Dec

volume: 18

issue: 4

pages: 173-8

abstract: PURPOSE: We evaluated the efficacy, safety and psychological aspect of monthly administrations of the gonadotropin-releasing hormone agonists (GnRHa), leuprolide acetate depot (Luphere depot 3.75 mg), in patients with precocious puberty. METHODS: A total of 54 girls with central precocious puberty were administered with leuprolide acetate (Luphere depot 3.75 mg) every four weeks over 24 weeks. We evaluated the percentage of children exhibiting a suppressed luteinizing hormone (LH) response to GnRH (LH peak≤3 IU/L), peak LH/follicle stimulating hormone (FSH) ratio of GnRH stimulation test less than 1, change in bone age/chronologic age ratio, change in the Tanner stage and change in eating habit and psychological aspect. RESULTS: (1) The percentage of children exhibiting a suppressed LH response to GnRH, defined as an LH peak≤3 IU/L at 24 weeks was 96.3 % (52/54). (2) The percentage of children exhibiting peak LH/FSH ratio<1 at 24 weeks of the study was 94.4 % (51/54). (3) The ratio of bone age and chronological age significantly declined from 1.27±0.07 to 1.24±0.01 after the 6 months of the study. (4) The mean Tanner stage manifested a significant change 2.3±0.48 at baseline, down to 1.70±0.61 at 24 weeks. (5) Based on the questionnaires, the score for eating habits showed a significant change from the baseline 34.0±6.8 to 31.3±6.8. (6) The psychological assessment did not exhibit a significant difference except with scores for sociability, problem behavior total score and other problems. CONCLUSION: The leuprolide 3.75 mg (Luphere depot) is useful and safety for treating children with central precocious puberty.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4027087/

Record 1348

pmcid: PMC4184537

pmid: 25356204

doi: 10.1002/ccr3.7

title: Multiple bone metastases detected 10 years after mastectomy with silicone reconstruction for DCIS and contralateral augmentation.

authors

- Mori R

- Nagao Y

journal: Clinical case reports

journalAbbr: Clin Case Rep

year: 2013

pubdate: 2013 Oct

volume: 1

issue: 1

pages: 26-9

abstract: The patient developed multiple bone metastases following mastectomy with silicone reconstruction and contralateral augmentation for ductal carcinoma in situ (DCIS) of the breast. She was diagnosed with contralateral invasive cancer. Distant metastasis of DCIS is rare, and other metastatic origins must be screened. However, screening of augmented breasts is difficult.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4184537/

Record 1349

pmcid: PMC4422278

pmid: 26023451

doi:

title: Neonatal mastauxe (breast enlargement of the newborn).

authors

- Raveenthiran V

journal: Journal of neonatal surgery

journalAbbr: J Neonatal Surg

year: 2013

pubdate: 2013 Jul-Sep

volume: 2

issue: 3

pages: 31

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4422278/

Record 1350

pmcid: PMC3782962

pmid: 24086905

doi: 10.7860/JCDR/2013/5503.3257

title: Parameatal cyst: a presentation of rare case and review of literature.

authors

- S L
- Ankur A

journal: Journal of clinical and diagnostic research : JCDR

journalAbbr: J Clin Diagn Res

year: 2013

pubdate: 2013 Aug

volume: 7

issue: 8

pages: 1757-8

abstract: A parameatal urethral cyst is a very rare congenital anomaly. It was first reported in two males in 1956 by Thompson and Lantin. About 50 cases have been published since then. Most of the cases which have been reported were from Japanese population and on extensive literature search, few cases were found to have been reported from India. We are reporting a case of a parameatal urethral cyst in a 7-year-old boy. Complete excision of the cyst with total removal of the epithelium is required for treatment and for prevention of recurrence.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3782962/

Record 1351

pmcid: PMC3875252

pmid: 24391378

doi:

title: Pharmaceutical approval update.

authors

- Goldenberg MM

journal: P & T : a peer-reviewed journal for formulary management

journalAbbr: P T

year: 2013

pubdate: 2013 Oct

volume: 38

issue: 10

pages: 603-36

abstract: Dolutegravir (Tivicay) for HIV-1 infection; topiramate extended release (Trokendi XR) for epilepsy; and ferric carboxymaltose (Injectafer) for iron-deficiency anemia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3875252/

Record 1352

pmcid: PMC3728636

pmid: 24415974

doi: 10.1159/000352098

title: Pneumomastia: a rare cause of breast swelling.

authors

- Kipling M
- Christou C
- Barthelmes L

journal: Breast care (Basel, Switzerland)

journalAbbr: Breast Care (Basel)

year: 2013

pubdate: 2013 Jun

volume: 8

issue: 3

pages: 218-9

abstract: BACKGROUND: Pneumomastia is air within the breast parenchyma. A number of causes have been reported for this condition. This case report describes a new cause and details of the management strategy applied, together with a review of the literature. CASE REPORT: We describe a case of acute breast swelling in a 40-year-old woman and its subsequent successful conservative management. CONCLUSION: Bronchopleural fistula after thoracotomy is a risk, and can cause pneumomastia. This is more likely to occur after redo thoracic surgery. Pneumomastia after repeat thoracotomy can be managed conservatively, even in the presence of a bronchopleural fistula.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3728636/

Record 1353

pmcid: PMC3694125

pmid: 23840581

doi: 10.1371/journal.pone.0067035

title: Polymorphisms of Pyrimidine Pathway Enzymes Encoding Genes and HLA-B*40:01 Carriage in Stavudine-Associated Lipodystrophy in HIV-Infected Patients.

authors

- Domingo P
- Mateo MG
- Pruvost A
- Torres F
- Salazar J
- Gutierrez Mdel M
- Domingo JC
- Fernandez I
- Villarroya F
- Vidal F
- Baiget M
- de la Calle-Martín O

journal: PloS one

journalAbbr: PLoS One

year: 2013

pubdate: 2013

volume: 8

issue: 6

pages: e67035

abstract: PURPOSE: To assess in a cohort of Caucasian patients exposed to stavudine (d4T) the association of polymorphisms in pyrimidine pathway enzymes and HLA-B*40:01 carriage with HIV/Highly active antiretroviral therapy (HAART)-associated lipodystrophy syndrome (HALS). METHODS: Three-hundred and thirty-six

patients, 187 with HALS and 149 without HALS, and 72 uninfected subjects were recruited. The diagnosis of HALS was performed following the criteria of the Lipodystrophy Severity Grading Scale. Polymorphisms in the thymidylate synthase (TS) and methylene-tetrahydrofolate reductase (MTHFR) genes were determined by direct sequencing, HLA-B genotyping by PCR-SSOr Luminex Technology, and intracellular levels of stavudine triphosphate (d4T-TP) by a LC-MS/MS assay method. RESULTS: HALS was associated with the presence of a low expression TS genotype polymorphism (64.7% vs. 42.9%, OR = 2.43; 95%CI: 1.53-3.88, P<0.0001). MTHFR gene polymorphisms and HLA-B*40:01 carriage were not associated with HALS or d4T-TP intracellular levels. Low and high expression TS polymorphisms had different d4T-TP intracellular levels (25.60 vs. 13.60 fmol/10(6) cells, P<0.0001). Independent factors associated with HALS were(OR [95%CI]: (a) Combined TS and MTHFR genotypes (p = 0.006, reference category (ref.): 'A+A'; OR for 'A+B' vs. ref.: 1.39 [0.69-2.80]; OR for 'B+A' vs. ref.: 2.16 [1.22-3.83]; OR for 'B+B' vs. ref.: 3.13, 95%CI: 1.54-6.35), (b) maximum viral load ≥5 log10 (OR: 2.55, 95%CI: 1.56-4.14, P = 0.001), (c) use of EFV (1.10 [1.00-1.21], P = 0.008, per year of use). CONCLUSION: HALS is associated with combined low-expression TS and MTHFR associated with high activity polymorphisms but not with HLA-B*40:01 carriage in Caucasian patients with long-term exposure to stavudine.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3694125/

Record 1354

pmcid: PMC3644938

pmid: 23632173

doi: 10.1136/bcr-2012-008455

title: Poorly controlled type 2 diabetes complicated by an episode of severe hypertriglyceridaemia-induced pancreatitis.

authors

- Denecker N
- Decochez K

journal: BMJ case reports

journalAbbr: BMJ Case Rep

year: 2013

pubdate: 2013 Apr 29

volume: 2013

issue:

pages:

abstract: A 23-year-old woman with a history of type 2 diabetes and non-compliance presented to the emergency department with abdominal epigastric pain and nausea. Laboratory examination revealed a mild ketoacidosis while an abdominal CT scan performed the following day demonstrated a severe acute pancreatitis of the body and tail (Balthazar grade E) despite normal amylase serum levels on admission. The presence of a lactescent serum was the clue to an extremely high triglyceride level (>10 000 mg/dl) causing the pancreatitis. The hypertriglyceridaemia itself was attributed mainly to the diabetic ketoacidosis. There was no family history of hypertriglyceridaemia. The triad consisting of diabetic ketoacidosis, hypertriglyceridaemia and acute pancreatitis is an unusual presentation of poorly controlled diabetes which can occur in type 1 as well as type 2 diabetic adults and children. Treatment with intravenous insulin and hydration successfully resolved the ketoacidosis and hypertriglyceridaemia and reversed the episode of acute pancreatitis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3644938/

Record 1355

pmcid: PMC3850124

pmid:

doi: 10.1186/1687-9856-2013-S1-P80

title: Precocious puberty and ovarian tumors – 2 case reports.

authors

- Ramkumar S
- Shivprakash
- Ammini AC

journal: International Journal of Pediatric Endocrinology

journalAbbr: Int J Pediatr Endocrinol

year: 2013

pubdate: 2013

volume: 2013

issue: Suppl 1

pages: P80

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3850124/

Record 1356

pmcid: PMC3850123

pmid:

doi: 10.1186/1687-9856-2013-S1-P78

title: Pseudo-precocious puberty in a 3-year old girl.

authors

- Pangilinan-Vazquez CAG
- Cua SC
- Abad LR
- Domingo CF
- Estrada SC
- Santos CM

journal: International Journal of Pediatric Endocrinology

journalAbbr: Int J Pediatr Endocrinol

year: 2013

pubdate: 2013

volume: 2013

issue: Suppl 1

pages: P78

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3850123/

Record 1357

pmcid: PMC7120621

pmid:

doi: 10.1007/978-3-642-34147-2_9

title: Pulmonary Interstitium.

authors

- Bayona JC
- Álvarez LA
- León PO

journal: Learning Chest Imaging

journalAbbr: Learning Chest Imaging

year: 2013

pubdate: 2013

volume:

issue:

pages: 197-219

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7120621/

Record 1358

pmcid: PMC3578540

pmid: 24426698

doi: 10.1007/s13193-012-0201-z

title: Recent advances in granulosa cell tumor ovary: a review.

authors

- Kottarathil VD
- Antony MA
- Nair IR
- Pavithran K

journal: Indian journal of surgical oncology

journalAbbr: Indian J Surg Oncol

year: 2013

pubdate: 2013 Mar

volume: 4

issue: 1

pages: 37-47

abstract: Granulosa cell tumors constitute less than 5 % of all ovarian tumors. Unlike epithelial ovarian tumors, they occur in a younger age group, are usually detected in an early stage and often have features of hyperestrogenism. The presenting symptoms are usually nonspecific with abdominal pain or distension. They follow an indolent course and are characterized by a long natural history. Mutation of FOXL2 (402C->G) seen in 97 % of adult GCT may be pathognomonic for adult GCT. Only stage of the disease has been consistently shown in various studies to affect survival of patients with GCT. The initial management of patients, for whom fertility is not an issue, is total abdominal hysterectomy, bilateral salpingo-oophorectomy and removal of all gross disease. Nodal dissection is not a significant factor for survival and is not recommended in surgical staging of GCT. Fertility preserving surgery with unilateral salpingo-oophorectomy is feasible in young patients

with stage Ia GCT. Patients with early stage disease (stage I and II) have a very good prognosis with 5 year DFS and OS of 89 % and 99 % respectively and these groups of patients usually don't require any postoperative treatment. Patients with stage Ic disease associated with poor prognostic factors like large tumor size or high mitotic index and stage II, have a higher chance of relapse, and may benefit with postoperative treatment but role of chemotherapy is still debatable. In advanced stage disease (stage III and IV) the 5 year DFS and OS disease was 72 % and 80 % respectively hence the option of postoperative treatment with 6 cycles of BEP should be considered in this group. Recently paclitaxel is being investigated as an effective tool in GCT. The efficacy of radiation in GCT is not well defined but in optimally debulked cases postoperative radiation is a viable option. Due to high chance of recurrence even years after apparent clinical cure of the primary tumor, lifelong follow up with clinical examination and tumor markers like inhibin B is recommended. About 25 % GCT develop recurrence and the median time to recur is usually 4-5 years. Most recurrences are intraperitoneal and usually a complete debulking of the disease is feasible even in the recurrent setting. Postoperative chemotherapy (platinum based) is usually given after surgery more so in cases with widespread disease or after suboptimal cytoreduction. Recurrent chemoresistant, progressive non-responding GCT or patients with high surgical risk are ideal candidates for targeted therapy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3578540/

Record 1359

pmcid: PMC3625151

pmid: 23593417

doi: 10.1371/journal.pone.0061160

title: Recombinant human growth hormone and rosiglitazone for abdominal fat accumulation in HIV-infected patients with insulin resistance: a randomized, double-blind, placebo-controlled, factorial trial.

authors

- Glesby MJ
- Albu J
- Chiu YL
- Ham K
- Engelson E
- He Q
- Muthukrishnan V
- Ginsberg HN
- Donovan D
- Ernst J
- Lesser M
- Kotler DP

journal: PloS one

journalAbbr: PLoS One

year: 2013

pubdate: 2013

volume: 8

issue: 4

pages: e61160

abstract: BACKGROUND: Recombinant human growth hormone (rhGH) reduces visceral adipose tissue (VAT) volume in HIV-infected patients but can worsen glucose homeostasis and lipoatrophy. We aimed to determine if adding rosiglitazone to rhGH would abrogate the adverse effects of rhGH on insulin sensitivity (SI) and

subcutaneous adipose tissue (SAT) volume. METHODOLOGY/PRINCIPAL FINDINGS: Randomized, double-blind, placebo-controlled, multicenter trial using a 2×2 factorial design in which HIV-infected subjects with abdominal obesity and insulin resistance were randomized to rhGH 3 mg daily, rosiglitazone 4 mg twice daily, combination rhGH + rosiglitazone, or double placebo (control) for 12 weeks. The primary endpoint was change in SI by frequently sampled intravenous glucose tolerance test from entry to week 12. Body composition was assessed by whole body magnetic resonance imaging (MRI) and dual Xray absorptiometry (DEXA). Seventy-seven subjects were randomized of whom 72 initiated study drugs. Change in SI from entry to week 12 differed across the 4 arms by 1-way ANCOVA (P = 0.02); by pair-wise comparisons, only rhGH (decreasing SI; P = 0.03) differed significantly from control. Changes from entry to week 12 in fasting glucose and glucose area under the curve on 2-hour oral glucose tolerance test differed across arms (1-way ANCOVA P = 0.004), increasing in the rhGH arm relative to control. VAT decreased significantly in the rhGH arms (-17.5% in rhGH/rosiglitazone and -22.7% in rhGH) but not in the rosiglitazone alone (-2.5%) or control arms (-1.9%). SAT did not change significantly in any arm. DEXA results were consistent with the MRI data. There was no significant rhGH x rosiglitazone interaction for any body composition parameter. CONCLUSIONS/SIGNIFICANCE: The addition of rosiglitazone abrogated the adverse effects of rhGH on insulin sensitivity and glucose tolerance while not significantly modifying the lowering effect of rhGH on VAT. TRIAL REGISTRATION: Clinicaltrials.gov NCT00130286.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3625151/

Record 1360

pmcid: PMC3590195

pmid: 23483919

doi: 10.1371/journal.pone.0057623

title: Resource security impacts men's female breast size preferences.

authors

- Swami V
- Tovée MJ

journal: PloS one

journalAbbr: PLoS One

year: 2013

pubdate: 2013

volume: 8

issue: 3

pages: e57623

abstract: It has been suggested human female breast size may act as signal of fat reserves, which in turn indicates access to resources. Based on this perspective, two studies were conducted to test the hypothesis that men experiencing relative resource insecurity should perceive larger breast size as more physically attractive than men experiencing resource security. In Study 1, 266 men from three sites in Malaysia varying in relative socioeconomic status (high to low) rated a series of animated figures varying in breast size for physical attractiveness. Results showed that men from the low socioeconomic context rated larger breasts as more attractive than did men from the medium socioeconomic context, who in turn perceived larger breasts as attractive than men from a high socioeconomic context. Study 2 compared the breast size judgements of 66 hungry versus 58 satiated men within the same environmental context in Britain. Results showed that hungry men rated larger breasts as significantly more attractive than satiated men. Taken together, these studies provide evidence that resource security impacts upon men's attractiveness ratings based on women's breast size.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3590195/

Record 1361

pmcid: PMC3846723

pmid: 24312671

doi: 10.1371/journal.pone.0082749

title: Serum prolactin and macroprolactin levels among outpatients with major depressive disorder following the administration of selective serotonin-reuptake inhibitors: a cross-sectional pilot study.

authors

- Kim S
- Park YM

journal: PloS one

journalAbbr: PLoS One

year: 2013

pubdate: 2013

volume: 8

issue: 12

pages: e82749

abstract: Clinical trials evaluating the rate of short-term selective serotonin-reuptake inhibitor (SSRI)-induced hyperprolactinemia have produced conflicting results. Thus, the aim of this study was to clarify whether SSRI therapy can induce hyperprolactinemia and macroprolactinemia. Fifty-five patients with major depressive disorder (MDD) were enrolled in this study. Serum prolactin and macroprolactin levels were measured at a single time point (i.e., in a cross-sectional design). All patients had received SSRI monotherapy (escitalopram, paroxetine, or sertraline) for a mean of 14.75 months. Their mean prolactin level was 15.26 ng/ml. The prevalence of patients with hyperprolactinemia was 10.9% for 6/55, while that of patients with macroprolactinemia was 3.6% for 2/55. The mean prolactin levels were 51.36 and 10.84 ng/ml among those with hyperprolactinemia and a normal prolactin level, respectively. The prolactin level and prevalence of hyperprolactinemia did not differ significantly within each SSRI group. Correlation analysis revealed that there was no correlation between the dosage of each SSRI and prolactin level. These findings suggest that SSRI therapy can induce hyperprolactinemia in patients with MDD. Clinicians should measure and monitor serum prolactin levels, even when both SSRIs and antipsychotics are administered.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3846723/

Record 1362

pmcid: PMC4579676

pmid: 26673282

doi: 10.15557/JoU.2013.0044

title: Sonography of abdominal organs in precocious puberty in girls.

authors

- Mazgaj M

journal: Journal of ultrasonography

journalAbbr: J Ultrason

year: 2013

pubdate: 2013 Dec

volume: 13

issue: 55

pages: 418-24

abstract: Precocious puberty constitutes a significant clinical problem due to psychological implications and health concerns as well as consequences associated with girls' fertility. Self-acceptance, peer approval, early motherhood and future fertility - these are only a few issues associated with puberty, the disorders of which may have a negative influence on personality and health. The role of imaging is to determine the causes of early activation of the puberty process, to identify lesions responsible for abnormal sex hormone production and those which are the result of underlying hormonal disorders as well as to diagnose the processes which only mimic symptoms of precocious puberty. Out of all available imaging methods, sonography, thanks to its safety, availability and low cost, seems to be the best method to assess the breasts and internal organs of patients manifesting symptoms of precocious puberty both in early diagnosis and follow-up examinations. Apart from the technique of performing ultrasound examinations, the paper also explains its role in precocious puberty in girls, describes correct parameters of the organs which play the most significant role in the process of puberty as well as presents the features of changes responsible for or resulting from hormonal disorders.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4579676/

Record 1363

pmcid: PMC3881144

pmid: 24322394

doi: 10.3390/ijerph10126832

title: Spatial distribution of cosmetic-procedure businesses in two U.S. cities: a pilot mapping and validation study.

authors

- Austin SB
- Gordon AR
- Kennedy GA
- Sonneville KR
- Blossom J
- Blood EA

journal: International journal of environmental research and public health

journalAbbr: Int J Environ Res Public Health

year: 2013

pubdate: 2013 Dec 6

volume: 10

issue: 12

pages: 6832-62

abstract: Cosmetic procedures have proliferated rapidly over the past few decades, with over $11 billion spent on cosmetic surgeries and other minimally invasive procedures and another $2.9 billion spent on U.V. indoor tanning in 2012 in the United States alone. While research interest is increasing in tandem with the growth of the industry, methods have yet to be developed to identify and geographically locate the myriad types of businesses purveying cosmetic procedures. Geographic location of cosmetic-procedure businesses is a critical element in understanding the public health impact of this industry; however no studies we are aware of have developed valid and feasible methods for spatial analyses of these types of businesses. The aim of this pilot validation study was to establish the feasibility of identifying businesses offering surgical and minimally invasive cosmetic procedures and to characterize the spatial distribution of these businesses. We developed and tested three methods for creating a geocoded list of cosmetic-procedure businesses in Boston (MA) and Seattle (WA), USA, comparing each method on sensitivity and staff time required per confirmed cosmetic-procedure business. Methods varied substantially. Our findings represent an important step toward enabling rigorous health-linked

spatial analyses of the health implications of this little-understood industry.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3881144/

Record 1364

pmcid: PMC3825644

pmid: 24132407

doi: 10.1007/s12630-013-0019-3

title: The difficult airway with recommendations for management--part 1--difficult tracheal intubation encountered in an unconscious/induced patient.

authors

- Law JA
- Broemling N
- Cooper RM
- Drolet P
- Duggan LV
- Griesdale DE
- Hung OR
- Jones PM
- Kovacs G
- Massey S
- Morris IR
- Mullen T
- Murphy MF
- Preston R
- Naik VN
- Scott J
- Stacey S
- Turkstra TP
- Wong DT

journal: Canadian journal of anaesthesia = Journal canadien d'anesthesie

journalAbbr: Can J Anaesth

year: 2013

pubdate: 2013 Nov

volume: 60

issue: 11

pages: 1089-118

abstract: BACKGROUND: Previously active in the mid-1990s, the Canadian Airway Focus Group (CAFG) studied the unanticipated difficult airway and made recommendations on management in a 1998 publication. The CAFG has since reconvened to examine more recent scientific literature on airway management. The Focus Group's mandate for this article was to arrive at updated practice recommendations for management of the unconscious/induced patient in whom difficult or failed tracheal intubation is encountered. METHODS: Nineteen clinicians with backgrounds in anesthesia, emergency medicine, and intensive care joined this iteration of the CAFG. Each member was assigned topics and conducted reviews of Medline, EMBASE, and Cochrane databases. Results were presented and discussed during multiple teleconferences and two face-to-face

meetings. When appropriate, evidence- or consensus-based recommendations were made together with assigned levels of evidence modelled after previously published criteria. CONCLUSIONS: The clinician must be aware of the potential for harm to the patient that can occur with multiple attempts at tracheal intubation. This likelihood can be minimized by moving early from an unsuccessful primary intubation technique to an alternative "Plan B" technique if oxygenation by face mask or ventilation using a supraglottic device is non-problematic. Irrespective of the technique(s) used, failure to achieve successful tracheal intubation in a maximum of three attempts defines failed tracheal intubation and signals the need to engage an exit strategy. Failure to oxygenate by face mask or supraglottic device ventilation occurring in conjunction with failed tracheal intubation defines a failed oxygenation, "cannot intubate, cannot oxygenate" situation. Cricothyrotomy must then be undertaken without delay, although if not already tried, an expedited and concurrent attempt can be made to place a supraglottic device.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3825644/

Record 1365

pmcid: PMC4174157

pmid: 25289234

doi: 10.1097/GOX.0b013e3182a7e827

title: The fate of nonvascularized fat grafts: histological and bioluminescent study.

authors

- Sunaga A
- Sugawara Y
- Katsuragi-Tomioka Y
- Kobayashi E

journal: Plastic and reconstructive surgery. Global open

journalAbbr: Plast Reconstr Surg Glob Open

year: 2013

pubdate: 2013 Sep

volume: 1

issue: 6

pages: e40

abstract: BACKGROUND: Nonvascularized fat grafting has become one of the most popular options for breast contouring. However, the survival process of the grafts remains to be elucidated. In this study, we tracked the fate of nonvascularized fat grafts with in vivo bioluminescence and immunohistochemistry. METHODS: Nonvascularized fat grafts or vascularized adiposal flaps from luciferase transgenic rats were transplanted to Lewis rats. The bioluminescent signals from the grafts were monitored longitudinally. In addition, nonvascularized fat grafts from Lewis rats were engrafted to Lewis rats and the viability of the adipocytes in the grafts was evaluated with immunohistochemical staining for perilipin at postoperative week 1, 2, 3, 4, and 6. RESULTS: The bioluminescent signals from the nonvascularized fat grafts increased drastically from postoperative day 3 to 7, stayed flat from day 7 to 12, and declined from day 12 to 17, whereas those from the vascularized fat flaps remained throughout the entire postoperative period. Immunohistochemistry revealed that the survival zones with large adipocytes were decreased within 2 weeks and the regenerating zones with small adipocytes appeared after 3 weeks. CONCLUSIONS: Our study showed the process of survival and regeneration of nonvascularized fat grafts and suggested that graft-derived stromal cells proliferated within 7 days after transplantation and differentiated into adipocytes after postoperative week 3.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4174157/

Record 1366

pmcid: PMC3998714

pmid: 23348055

doi: 10.1016/j.reprotox.2013.01.002

title: The male mammary gland: a target for the xenoestrogen bisphenol A.

authors

- Vandenberg LN
- Schaeberle CM
- Rubin BS
- Sonnenschein C
- Soto AM

journal: Reproductive toxicology (Elmsford, N.Y.)

journalAbbr: Reprod Toxicol

year: 2013

pubdate: 2013 Jun

volume: 37

issue:

pages: 15-23

abstract: Males of some strains of mice retain their mammary epithelium even in the absence of nipples. Here, we have characterized the mammary gland in male CD-1 mice both in whole mounts and histological sections. We also examined the effects of bisphenol A (BPA), an estrogen mimic that alters development of the female mouse mammary gland. BPA was administered at a range of environmentally relevant doses (0.25-250μg/kg/day) to pregnant and lactating mice and then the mammary glands of male offspring were examined at several periods in adulthood. We observed age- and dose-specific effects on mammary gland morphology, indicating that perinatal BPA exposures alter the male mammary gland in adulthood. These results may provide insight into gynecomastia, the most common male breast disease in humans, where proliferation of the mammary epithelium leads to breast enlargement.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3998714/

Record 1367

pmcid: PMC4400691

pmid: 25893059

doi:

title: The man with bilateral nipple pain.

authors

- Hong HC
- Koh KC

journal: Malaysian family physician : the official journal of the Academy of Family Physicians of Malaysia

journalAbbr: Malays Fam Physician

year: 2013

pubdate: 2013

volume: 8

issue: 3

pages: 43-5

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4400691/

Record 1368

pmcid: PMC3766216

pmid: 23968123

doi: 10.1186/1471-244X-13-214

title: Treating symptomatic hyperprolactinemia in women with schizophrenia: presentation of the ongoing DAAMSEL clinical trial (Dopamine partial Agonist, Aripiprazole, for the Management of Symptomatic ELevated prolactin).

authors

- Kelly DL
- Wehring HJ
- Earl AK
- Sullivan KM
- Dickerson FB
- Feldman S
- McMahon RP
- Buchanan RW
- Warfel D
- Keller WR
- Fischer BA
- Shim JC

journal: BMC psychiatry

journalAbbr: BMC Psychiatry

year: 2013

pubdate: 2013 Aug 22

volume: 13

issue:

pages: 214

abstract: Prolactin elevations occur in people treated with antipsychotic medications and are often much higher in women than in men. Hyperprolactinemia is known to cause amenorrhea, oligomenorrhea, galactorrhea and gynecomastia in females and is also associated with sexual dysfunction and bone loss. These side effects increase risk of antipsychotic nonadherence and suicide and pose significant problems in the long term management of women with schizophrenia. In this manuscript, we review the literature on prolactin; its physiology, plasma levels, side effects and strategies for treatment. We also present the rationale and protocol for an ongoing clinical trial to treat symptomatic hyperprolactinemia in premenopausal women with schizophrenia. More attention and focus are needed to address these significant side effects and help the field better personalize the treatment of women with schizophrenia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3766216/

Record 1369

pmcid: PMC3706051

pmid: 24872736

doi: 10.1055/s-0033-1343993

title: Uncommon benign breast abnormalities in adolescents.

authors

- Warren R
- Degnim AC

journal: Seminars in plastic surgery

journalAbbr: Semin Plast Surg

year: 2013

pubdate: 2013 Feb

volume: 27

issue: 1

pages: 26-8

abstract: The authors discuss benign breast abnormalities in the adolescent breast other than fibroadenoma. Although fibroadenoma is the most common benign abnormality in the adolescent breast, other diagnoses are possible. The majority of adolescents who present with a palpable concern or lump have no discrete abnormality on ultrasound and are diagnosed with clinical fibrocystic change and followed up to ensure clinical stability. Intraductal papilloma and duct ectasia are two benign abnormalities associated with bloody nipple discharge, occurring more rarely in adolescents compared with adult women. Breast infections can occur in adolescents, including both mastitis and/or abscess, and are treated similarly to adults, with drainage and antibiotic coverage for Staphylococcus. When infections are due to nipple piercing, other organisms should be suspected. All surgical procedures in the developing breast should be performed cautiously, as trauma to the undeveloped breast can result in failure of breast development or asymmetry, and surgical disruption of subareolar ducts can impair or preclude future lactation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3706051/

Record 1370

pmcid: PMC3808755

pmid: 24154518

doi: 10.1136/bmjopen-2013-003709

title: Use of autologous fat grafting for reconstruction postmastectomy and breast conserving surgery: a systematic review protocol.

authors

- Agha RA
- Goodacre T
- Orgill DP

journal: BMJ open

journalAbbr: BMJ Open

year: 2013

pubdate: 2013 Oct 22

volume: 3

issue: 10

pages: e003709

abstract: INTRODUCTION: There is growing interest in the potential use of autologous fat grafting (AFG) for the

purposes of breast reconstruction. However, concerns have been raised regarding the technique's clinical effectiveness, safety and interference with screening mammography. The objective of this systematic review was to determine the oncological, clinical, aesthetic and functional, patient reported, process and radiological outcomes for AFG. METHODS AND ANALYSIS: All original studies, including randomised controlled trials, cohorts studies, case-control studies, case series and case reports involving women undergoing breast reconstruction. All AFG techniques performed for the purposes of reconstruction in the postmastectomy or breast conserving surgery setting will be considered. Outcomes are defined within this protocol along; oncological, clinical, aesthetic and functional, patient reported, process and radiological domains. The search strategy has been devised to find papers about 'fat grafting and breast reconstruction' and is outlined within the body of this protocol. The full search strategy is outlined within the body of the protocol. The following electronic databases will be searched from 1 January 1986 to 6 June 2013: PubMed, MEDLINE, EMBASE, SCOPUS, CINAHL, PsycINFO, SciELO, The Cochrane Library, including the Cochrane Database of Systematic Reviews (CDSR), Cochrane Central Register of Controlled Trials (CENTRAL), Database of Abstracts of Reviews of Effect (DARE), the Cochrane Methodology Register, Health Technology Assessment Database, the NHS Economic Evaluation Databases and Cochrane Groups, ClinicalTrials.gov, Current Controlled Trials Database, the World Health Organisation (WHO) International Clinical Trials Registry Platform, UpToDate.com, NHS Evidence and the York Centre for Reviews and Dissemination. Grey literature searches will also be conducted as detailed in our review protocol. Eligibility assessment occurred in two stages, title and abstract screening and then full text assessment. Data were extracted and stored in a database with standardised extraction fields to facilitate easy and consistent data entry. ETHICS AND DISSEMINATION: This systematic review will be published in a peer-reviewed journal. It will also be presented at national and international conferences in the fields of plastic, reconstructive and aesthetic surgery and at more general surgical and methodological conferences. It will be disseminated electronically and in print. Brief reports of the review findings will be disseminated directly to the appropriate audiences of surgeons and societies through email and other modes of communication. Updates of the review will be conducted to inform and guide healthcare practice and policy. PROTOCOL REGISTRATION: PROSPERO-National Institute of Health Research (NIHR) Prospective Register of Systematic Reviews (CRD42013005254).

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3808755/

Record 1371

pmcid: PMC4027073

pmid: 24904863

doi: 10.6065/apem.2013.18.3.111

title: Various endocrine disorders in children with t(13;14)(q10;q10) Robertsonian translocation.

authors

- Choi BH
- Kim UH
- Lee KS
- Ko CW

journal: Annals of pediatric endocrinology & metabolism

journalAbbr: Ann Pediatr Endocrinol Metab

year: 2013

pubdate: 2013 Sep

volume: 18

issue: 3

pages: 111-5

abstract: PURPOSE: 45,XY,t(13;14)(q10;q10) karyotype can suggest infertility associated with more or less severe oligospermia in male adults. In addition, 45,XX,t(13;14)(q10;q10) karyotype carries reproductive risks

such as miscarriage or infertility in female adults. However, reports on the phenotype of this karyotype in children are very rare. This study was done to observe various phenotypes of this karyotype in children. METHODS: Between January 2007 and December 2012, children diagnosed with 45,XY,t(13;14)(q10;q10) or 45,XX,t(13;14)(q10;q10) karyotype by chromosome analysis were analyzed retrospectively. RESULTS: Eight children (5 boys and 3 girls) were diagnosed with 45,XY,t(13;14)(q10;q10) or 45,XX,t(13;14)(q10;q10) karyotype. They ranged in age from 5 years and 6 months to 12 years and 4 months. The phenotypes of the study patients consisted of 1 hypogonadotrophic hypogonadism, 1 precocious puberty, 3 early puberty, 2 growth hormone deficiency (GHD) (partial) and 1 idiopathic short stature. As shown here t(13;14)(q10;q10) Robertsonian translocation shows a wide range of phenotypes. CONCLUSION: It can be said that t(13;14)(q10;q10) Robertsonian translocation shows various phenotypes from GHD to precocious puberty in children. Further large-scale studies are necessary.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4027073/

Record 1372

pmcid: PMC3401151

pmid: 22826780

doi: 10.4081/rt.2012.e23

title: A case of a giant pseudoangiomatous stromal hyperplasia of the breast: magnetic resonance imaging findings.

authors

- Solomou E
- Kraniotis P
- Patriarcheas G

journal: Rare tumors

journalAbbr: Rare Tumors

year: 2012

pubdate: 2012 Apr 12

volume: 4

issue: 2

pages: e23

abstract: Pseudoangiomatous stromal hyperplasia (PASH) of the breast is a benign myofibroblastic process. We present the case of a 17-year-old girl who underwent diagnostic work-up due to an enlargement of her left breast. She was submitted to ultrasounds and magnetic resonance imaging (MRI) which depicted a 14 cm lesion in her left breast. The patient was later operated and histology revealed PASH. Although PASH may range from 0.6-12 cm, a few lesions over 12 cm have been described, the largest being 20 cm. Large series present mammographic and ultrasonographic features of PASH in the literature, but little has been reported on the MR characteristics of PASH up to today. Signal on the T1-weighted image (T1WI) and T2-weighted image (T2WI) may vary. Curves generated from dynamic contrast-enhanced magnetic resonance imaging (DCE-MRI) studies are mainly type I or less frequently type II. There are no reports about diffusion-weighted imaging and corresponding apparent diffusion coefficient (ADC) values for PASH in the literature. ADC values in our case lie within the range of values reported for other benign breast lesions. The presence of slit-like spaces within the lesion on MR imaging along with DCE-MRI type I curve and ADC values consistent with a benign lesion may favour the diagnosis of PASH. Tissue biopsy is necessary, however for the final diagnosis. This case report will further contribute to the understanding of MR imaging features of PASH, especially in cases where mammography is not indicated.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3401151/

Record 1373

pmcid: PMC3342101

pmid: 22433036

doi: 10.1186/1471-244X-12-20

title: A patient perspective of the impact of medication side effects on adherence: results of a cross-sectional nationwide survey of patients with schizophrenia.

authors

- Dibonaventura M
- Gabriel S
- Dupclay L
- Gupta S
- Kim E

journal: BMC psychiatry

journalAbbr: BMC Psychiatry

year: 2012

pubdate: 2012 Mar 20

volume: 12

issue:

pages: 20

abstract: BACKGROUND: Antipsychotic medications often have a variety of side effects, however, it is not well understood how the presence of specific side effects correlate with adherence in a real-world setting. The aim of the current study was to examine the relationship between these variables among community-dwelling patients with schizophrenia. METHODS: Data were analyzed from a 2007-2008 nationwide survey of adults who self-reported a diagnosis of schizophrenia and were currently using an antipsychotic medication (N = 876). The presence of side effects was defined as those in which the patient reported they were at least "somewhat bothered". Adherence was defined as a score of zero on the Morisky Medication Adherence Scale. To assess the relationship between side effects and adherence, individual logistic regression models were fitted for each side effect controlling for patient characteristics. A single logistic regression model assessed the relationship between side effect clusters and adherence. The relationships between adherence and health resource use were also examined. RESULTS: A majority of patients reported experiencing at least one side effect due to their medication (86.19%). Only 42.5% reported complete adherence. Most side effects were associated with a significantly reduced likelihood of adherence. When grouped as side effect clusters in a single model, extra pyramidal symptoms (EPS)/agitation (odds ratio (OR) = 0.57, p = 0.0007), sedation/cognition (OR = 0.70, p = 0.033), prolactin/endocrine (OR = 0.69, p = 0.0342), and metabolic side effects (OR = 0.64, p = 0.0079) were all significantly related with lower rates of adherence. Those who reported complete adherence to their medication were significantly less likely to report a hospitalization for a mental health reason (OR = 0.51, p = 0.0006), a hospitalization for a non-mental health reason (OR = 0.43, p = 0.0002), and an emergency room (ER) visit for a mental health reason (OR = 0.60, p = 0.008). CONCLUSIONS: Among patients with schizophrenia, medication side effects are highly prevalent and significantly associated with medication nonadherence. Nonadherence is significantly associated with increased healthcare resource use. Prevention, identification, and effective management of medication-induced side effects are important to maximize adherence and reduce health resource use in schizophrenia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3342101/

Record 1374

pmcid: PMC3534206

pmid: 23320206

doi: 10.1155/2012/529730

title: A rare corticotroph-secreting tumor with coexisting prolactin and growth hormone staining cells.

authors

- Kannan S
- Staugaitis SM
- Weil RJ
- Hatipoglu B

journal: Case reports in endocrinology

journalAbbr: Case Rep Endocrinol

year: 2012

pubdate: 2012

volume: 2012

issue:

pages: 529730

abstract: Pituitary adenomas can express and secrete different hormones. Expression of pituitary hormones in nonneoplastic pituitary cells is regulated by different transcription factors. Some pituitary adenomas show plurihormonal expression. The most commonly reported plurihormonal adenomas are composed of somatotrophs, lactotrophs, thyrotrophs and gonadotrophs. Pituitary adenomas composed of both corticotroph and somatolactotroph secreting cells are not common because transcription factors regulating the expression of these hormones are different. We report a rare case of pituitary adenoma with concomitant corticotroph, prolactin, and growth hormone staining cells, review literature on similar cases, and discuss possible biological mechanisms underlying these plurihormonal tumors.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3534206/

Record 1375

pmcid: PMC3678823

pmid: 23908849

doi: 10.5041/RMMJ.10089

title: Accelerated Aging in HIV Patients.

authors

- Meir-Shafrir K
- Pollack S

journal: Rambam Maimonides medical journal

journalAbbr: Rambam Maimonides Med J

year: 2012

pubdate: 2012 Oct

volume: 3

issue: 4

pages: e0025

abstract: Life expectancy has been increasing in the last few decades in the Western world and is accompanied by higher occurrence of age-related diseases like metabolic, cardiovascular, and renal diseases and also with a decline in immune functions. In HIV-infected people, due to the use of combination antiretroviral therapy (cART), life expectancy has increased. As a result, non-AIDS conditions which are age-associated have become more prevalent and appear earlier, resulting in accelerated aging in HIV patients. These non-AIDS conditions in HIV

patients are associated with CD4+ T cell counts: lower counts are associated with higher rates of liver, cardiovascular, renal, and neurocognitive disorders. The effect of viral load and cART on the earlier occurrence of age-associated diseases is less significant than the CD4 count effect. Thus, the loss of immune functions in HIV-infected patients may enhance aging.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3678823/

Record 1376

pmcid: PMC3412317

pmid: 22891078

doi: 10.3410/M4-16

title: Active surveillance for low-risk prostate cancer.

authors

- Klotz L

journal: F1000 medicine reports

journalAbbr: F1000 Med Rep

year: 2012

pubdate: 2012

volume: 4

issue:

pages: 16

abstract: Active surveillance for localized prostate cancer entails initial expectant management rather than immediate therapy, with "curative-intent" treatment deferred until there is evidence that the patient is at increased risk for disease progression. This is a response to the clearly documented risks of over diagnosis and overtreatment of low-risk prostate cancer, which in most cases poses little or no threat to the patient. It is based upon the prolonged natural history of prostate cancer and is an attempt to balance the risks and side effects of overtreatment against the possibility of disease progression and a lost opportunity for cure. Low-risk prostate cancer is more accurately viewed as one of multiple risk factors for the presence of higher grade prostate cancer. Like atypical small acinar proliferation, (ASAP), it may be managed with close follow up but without radical intervention, unless there is clear evidence of more aggressive disease.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3412317/

Record 1377

pmcid: PMC4344966

pmid: 25734044

doi:

title: Anaplastic large cell lymphoma associated with breast implant: a case report.

authors

- Rajabiani A
- Arab H
- Emami A
- Manafi A
- Bazzaz N
- Saffar H

journal: World journal of plastic surgery

journalAbbr: World J Plast Surg

year: 2012

pubdate: 2012 Jan

volume: 1

issue: 1

pages: 46-50

abstract: Primary breast lymphoma represents less than 1% of all primary breast malignancies and most primary breast lymphomas are of B-Cell origin. The association of anaplastic lymphoma kinase (ALK) negative anaplastic large cell lymphoma (ALCL), a very rare form of primary breast lymphoma, with silicone-filled breast implants has been suggested and several case reports supported this proposal, especially in Western countries. Here we describe one of the first cases of primary breast ALK-negative ALCL in association with saline-filled silicone breast implants evaluated in Iran, where the rising number of breast reconstructive and aesthetic surgeries would commit both surgical pathologists and plastic surgeons to be familiar with this entity.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4344966/

Record 1378

pmcid: PMC3372809

pmid: 22707897

doi: 10.3904/kjim.2012.27.2.224

title: Angiosarcomas of the bilateral breast and heart: which one is the primary site?

authors

- Kim EK
- Park IS
- Sohn BS
- Kim JE
- Lee DH
- Kim SW
- Suh C

journal: The Korean journal of internal medicine

journalAbbr: Korean J Intern Med

year: 2012

pubdate: 2012 Jun

volume: 27

issue: 2

pages: 224-8

abstract: A 29-year-old pregnant woman with recurrent pericardial effusion and a cardiac tumor, diagnosed as an angiosarcoma, was treated with surgical resection of the tumor followed by radiotherapy. Immediately after completion of radiotherapy, she developed bilateral breast masses, which were also confirmed as angiosarcomas. We thought this might be the first case of bilateral angiosarcoma of the breast metastasizing to heart mimicking a primary cardiac angiosarcoma, although we could not conclude with certainty that angiosarcoma of the heart was not the primary site.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3372809/

Record 1379

pmcid: PMC3357572

pmid: 22690360

doi: 10.4088/PCC.11l01239

title: Antipsychotic-induced euprolactinemic galactorrhea in an adolescent girl: a case report.

authors

- Kwak WJ
- Patel A
- Haq F
- Siddiqui F
- Younis M
- Raza S
- Gholkar R

journal: The primary care companion for CNS disorders

journalAbbr: Prim Care Companion CNS Disord

year: 2012

pubdate: 2012

volume: 14

issue: 1

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3357572/

Record 1380

pmcid: PMC3442184

pmid: 23024714

doi: 10.4066/AMJ.2012.1248

title: Antiretroviral therapy associated mastopathy.

authors

- Sankaye S
- Kachewar S

journal: The Australasian medical journal

journalAbbr: Australas Med J

year: 2012

pubdate: 2012

volume: 5

issue: 8

pages: 418-20

abstract: Reports about bilateral breast enlargement in patients on antiretroviral therapy are rare. It forms a constituent of the human immunodeficiency virus lipodystrophy syndrome. Often clinical suspicion followed by appropriate imaging evaluation is confirmative.A case of antiretroviral therapy associated mastopathy is therefore presented here so that increased awareness would obviate the need of mastectomy in such cases.

We also emphasise the role of adequate counselling in this scenario in alleviating patients' anxiety.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3442184/

Record 1381

pmcid: PMC3703173

pmid: 23853639

doi:

title: Bilateral breast swelling in a 23-year-old woman with Burkitt lymphoma.

authors

- Janbabai G
- Kayedimajd S
- Alian S
- Naghshvar F
- Rashidi M
- Farazmandfar T

journal: Journal of research in medical sciences : the official journal of Isfahan University of Medical Sciences

journalAbbr: J Res Med Sci

year: 2012

pubdate: 2012 Dec

volume: 17

issue: 12

pages: 1188-91

abstract: Primary breast lymphoma (PBL) of the breast is a rare type of localized non-Hodgkin lymphoma, which is usually of the B-cell. The majority of breast lymphoma present as a unilateral painless breast masses in an older woman, average age at diagnosis 55-60. A less common but distinctive presentation is a young woman of childbearing age who presents during or immediately after pregnancy. We present a 23-year-old postpartum woman with bilateral breast swelling. After surgical drainage and debridement and pathologic examination, the diagnosis of breast Burkitt lymphoma (BL) was confirmed. Chemotherapy was immediately started for her and the patient showed a good response with complete remission.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3703173/

Record 1382

pmcid: PMC3601661

pmid: 22484153

doi: 10.1016/j.steroids.2012.03.013

title: Biological characterization of non-steroidal progestins from botanicals used for women's health.

authors

- Toh MF
- Sohn J
- Chen SN
- Yao P
- Bolton JL
- Burdette JE

journal: Steroids

journalAbbr: Steroids

year: 2012

pubdate: 2012 Jun

volume: 77

issue: 7

pages: 765-73

abstract: Progesterone plays a central role in women's reproductive health. Synthetic progestins, such as medroxyprogesterone acetate (MPA) are often used in hormone replacement therapy (HRT), oral contraceptives, and for the treatment of endometriosis and infertility. Although MPA is clinically effective, it also promiscuously binds to androgen and glucocorticoid receptors (AR/GR) leading to many undesirable side effects including cardiovascular diseases and breast cancers. Therefore, identifying alternative progestins is clinically significant. The purpose of this study was to biologically characterize non-steroidal progestins from botanicals by investigating theirinteraction and activation of progesterone receptor (PR). Eight botanicals commonly used to alleviate menopausal symptoms were investigated to determine if they contain progestins using a progesterone responsive element (PRE) luciferase reporter assay and a PR polarization competitive binding assay. Red clover extract stimulated PRE-luciferase and bound to PR. A library of purified compounds previously isolated from red clover was screened using the luciferase reporter assay. Kaempferol identified in red clover and a structurally similar flavonoid, apigenin, bound to PR and induced progestegenic activity and P4 regulated genes in breast epithelial cells and human endometrial stromal cells (HESC). Kaempferol and apigenin demonstrated higher progestegenic potency in the HESC compared to breast epithelial cells. Furthermore, phytoprogestins were able to activate P4 signaling in breast epithelial cells without downregulating PR expression. These data suggest that botanical extracts used for women's health may contain compounds capable of activating progesterone receptor signaling.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3601661/

Record 1383

pmcid: PMC3433033

pmid: 22205436

doi: 10.1097/QAI.0b013e31824330cb

title: Body fat abnormality in HIV-infected children and adolescents living in Europe: prevalence and risk factors.

authors

- Alam N
- Cortina-Borja M
- Goetghebuer T
- Marczynska M
- Vigano A
- Thorne C

journal: Journal of acquired immune deficiency syndromes (1999)

journalAbbr: J Acquir Immune Defic Syndr

year: 2012

pubdate: 2012 Mar 1

volume: 59

issue: 3

pages: 314-24

abstract: OBJECTIVES: To estimate the prevalence of and identify risk factors for lipodystrophy syndrome (LS) and body fat abnormality in a population of HIV-infected children and adolescents. DESIGN: Cross-sectional observational study. METHODS: HIV-infected subjects aged 2-18 years were recruited from 15 HIV centers in Belgium, Italy, and Poland between January 2007 and December 2008. Standardized assessments by the patient's long-term clinician were performed to establish the presence of abnormality. Risk factors were explored in logistic regression models for fat abnormality outcomes and LS (abnormality plus dyslipidemia). RESULTS: Among 426 subjects (70% white), median age was 12.2 years (interquartile range: 9.0-15.0 years) and median duration of antiretroviral therapy was 5.2 years (interquartile range: 2.2-8.8 years). Prevalence was 57% (n = 235) for LS and 42% (n = 176) for fat abnormality; 90 subjects with abnormality were affected in ≥3 locations. Lipoatrophy occurred in 28% (n = 117) of subjects and lipohypertrophy in 27% (n = 115), most commonly in the face and trunk, respectively. In multivariable analysis, white ethnicity, body mass index, ritonavir/lopinavir, and nonnucleoside reverse transcriptase inhibitors were each associated with an increased risk of LS ($P < 0.05$). White ethnicity, history of Centers for Disease Control and Prevention-defined disease, and stavudine were associated with risk of lipoatrophy ($P < 0.05$). Increased risk of lipohypertrophy was associated with body mass index and prior HIV disease. CONCLUSIONS: Fat abnormality was prevalent in close to half of children and adolescents, who had accumulated long treatment durations. Risk of fat abnormality was associated with specific drugs, including stavudine and ritonavir, and other variables. Our results underline the importance of continued surveillance of children treated with antiretroviral therapy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3433033/

Record 1384

pmcid: PMC10426970

pmid: 22995446

doi: 10.1177/147470491201000319

title: Body image and body type preferences in St. Kitts, Caribbean: a cross- cultural comparison with U.S. samples regarding attitudes towards muscularity, body fat, and breast size.

authors

- Gray PB
- Frederick DA

journal: Evolutionary psychology : an international journal of evolutionary approaches to psychology and behavior

journalAbbr: Evol Psychol

year: 2012

pubdate: 2012 Sep 6

volume: 10

issue: 3

pages: 631-55

abstract: We investigated body image in St. Kitts, a Caribbean island where tourism, international media, and relatively high levels of body fat are common. Participants were men and women recruited from St. Kitts (n = 39) and, for comparison, U.S. samples from universities (n = 618) and the Internet (n = 438). Participants were shown computer generated images varying in apparent body fat level and muscularity or breast size and they indicated their body type preferences and attitudes. Overall, there were only modest differences in body type preferences between St. Kitts and the Internet sample, with the St. Kitts participants being somewhat more likely to value heavier women. Notably, however, men and women from St. Kitts were more likely to idealize smaller breasts than participants in the U.S. samples. Attitudes regarding muscularity were generally similar across samples. This study provides one of the few investigations of body preferences in the Caribbean.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10426970/

Record 1385

pmcid: PMC3332338

pmid: 22593626

doi: 10.1007/s00238-011-0595-9

title: Breast reshaping in gynecomastia by the "pull-through technique": considerations after 15 years.

authors

- Morselli PG
- Morellini A

journal: European journal of plastic surgery

journalAbbr: Eur J Plast Surg

year: 2012

pubdate: 2012 May

volume: 35

issue: 5

pages: 365-371

abstract: Gynecomastia corresponds to abnormal and excessive development of breast tissue in male patients. It may be unilateral or bilateral and, depending on breast volume and skin redundancy, it can be classified, according to Simon, in four different groups. Standard therapy is surgery and different techniques have been described so far, including different type of incisions in the periareolar or intrareolar skin. In 1996 in this journal we described our personal technique, called "pull-through." Our technique combines liposuction that is performed on two planes, subcutaneous and subglandular, and sharp parenchymal excision, performed through the small liposuction incisions. These incisions are short (1-1.5 cm) and hidden in the inframammary fold and behind the anterior axillary pillar. Furthermore the operation can be performed under local anesthesia and with a short recovery period. Since then we have operated on 260 patients. We now present our 15 years experience and our review of literature, focused on authors who used and criticized our technique. Results, in terms of breast volume reduction, skin retraction, and minimal visibility of scars, were very good, even in cases more severe than in our first series (Simon's type IIb and female-to-male transsexuals). Patients' satisfaction rate was high as well and the incidence of complications was low. The results we observed in our series and the ones presented by different authors confirmed the validity and feasibility of the "pull-through" technique in terms of esthetic and functional results, patient satisfaction, and a low incidence of complications.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3332338/

Record 1386

pmcid: PMC3533149

pmid: 23430037

doi:

title: Causes and types of precocious puberty in north-west iran.

authors

- Shiva S
- Fayyazi A
- Melikian A
- Shiva S

journal: Iranian journal of pediatrics

journalAbbr: Iran J Pediatr

year: 2012

pubdate: 2012 Dec

volume: 22

issue: 4

pages: 487-92

abstract: OBJECTIVE: Precocious puberty is of concern because of the underlying disorders, the short adult stature, and the psychosocial difficulties. This study was carried out in order to evaluate the characteristics of children referred to pediatric endocrinology clinic with diagnosis of precocious puberty. METHODS: In a cross-sectional study between February 2007 and September 2009, all of the children referred to pediatric endocrinology clinic in North-West Iran with diagnosis of precocious puberty were recruited. FINDINGS: Data of 106 girls (82.2%) and 23 boys (17.8%) were analyzed. Mean age of the patients at the time of referral was 6.6±2.8 years (ranging 0.3-14 yr), which was 7±3.9 (ranging 0.3-14 yr) for boys and 6.6±2.5 (ranging 0.8-12 yr) for girls (P=0.6). Out of 129 subjects, 56(43.4%) had precocious puberty, 71.4% (35 cases) of them were due to central precocious puberty and 28.6% (16 cases) were pseudo-precocious puberty. 73 out of 129 subjects (56.6%) were due to normal variants of puberty, normal puberty, and no puberty. 87.5% of subjects with central precocious puberty were idiopathic. CONCLUSION: Most of children referred with diagnosis of precocious puberty have benign normal variants. Most of cases with precocious puberty are affected with central precocious puberty, especially with idiopathic form of it.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3533149/

Record 1387

pmcid: PMC3401770

pmid: 22837930

doi: 10.4103/2230-8210.98027

title: Central precocious puberty due to hypothalamic hamartoma in a six-month-old infant girl.

authors

- Kotwal N
- Yanamandra U
- Menon AS
- Nair V

journal: Indian journal of endocrinology and metabolism

journalAbbr: Indian J Endocrinol Metab

year: 2012

pubdate: 2012 Jul

volume: 16

issue: 4

pages: 627-30

abstract: Precocious puberty defined as an onset of puberty below eight years in girls and nine years in boys, has an incidence of approximately 1 / 5,000 - 1 / 10,000 subjects with a female / male ratio of 20: 1. It is etiologically classified broadly as central and peripheral. We present to you a case of isosexual (central), precocious puberty in a 16-month-old girl, who was symptomatic since the age of six months, and was later, diagnosed to have hypothalamic hamartoma. It is one of the earliest case records ever in the medical literature of menarche, at an extremely early age (six-month-old child) secondary to a central cause.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3401770/

Record 1388

pmcid: PMC3460036

pmid: 22668795

doi: 10.1016/j.pneurobio.2012.05.012

title: Developing treatment for spinal and bulbar muscular atrophy.

authors

- Fischbeck KH

journal: Progress in neurobiology

journalAbbr: Prog Neurobiol

year: 2012

pubdate: 2012 Dec

volume: 99

issue: 3

pages: 257-61

abstract: Spinal and bulbar muscular atrophy is unique among the polyglutamine diseases in that the toxicity of the mutant protein, the androgen receptor, is ligand-dependent. In cell culture and animal models the mutant androgen receptor causes protein aggregation and alterations in transcriptional regulation, axonal transport, and mitochondrial function. Various therapeutic approaches have shown efficacy in mouse models, including androgen reduction and agents that alter the processing and degradation of the mutant androgen receptor protein, such as HSP90 inhibitors, IGF-1, and ASC-J9. Clinical trials of androgen-reducing agents have shown indications of efficacy but not proof of clinically meaningful benefit to date. This trial experience has set the stage for future clinical studies of other agents that have been found to be beneficial in transgenic animal models.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3460036/

Record 1389

pmcid: PMC3543238

pmid: 23272645

doi: 10.1186/1471-2407-12-626

title: Development and validation of a questionnaire to assess delay in treatment for breast cancer.

authors

- Unger-Saldaña K
- Peláez-Ballestas I
- Infante-Castañeda C

journal: BMC cancer

journalAbbr: BMC Cancer

year: 2012

pubdate: 2012 Dec 28

volume: 12

issue:

pages: 626

abstract: BACKGROUND: This study reports the reliability and validity of a questionnaire designed to measure the time from detection of a breast cancer to arrival at a cancer hospital, as well as the factors that are associated with delay. METHODS: The proposed questionnaire measures dates for estimation of the patient, provider and total intervals from detection to treatment, as well as factors that could be related to delays: means

of problem identification (self-discovery or screening), the patients' initial interpretations of symptoms, patients' perceptions of delay, reasons for delay in initial seeking of medical care, barriers perceived to have caused provider delay, prior utilisation of health services, use of alternative medicine, cancer-screening knowledge and practices, and aspects of the social network of support for medical attention. The questionnaire was assembled with consideration for previous research results from a review of the literature and qualitative interviews of patients with breast cancer symptoms. It was tested for face validity, content validity, reliability, internal consistency, convergent and divergent validity, sensitivity and specificity in a series of 4 tests with 602 patients. RESULTS: The instrument showed good face and content validity. It allowed discrimination of patients with different types and degrees of delay, had quite good reliability for the time intervals (with no significant mean differences between the two measurements), and fairly good internal consistency of the item dimensions (with Cronbach's alpha values for each dimension between 0.42 and 0.85). Finally, sensitivity and specificity were 74.68% and 48.81%, respectively. CONCLUSIONS: To the best of our knowledge, this is the first published report of the development and validation of a questionnaire for estimation of breast cancer delay and its correlated factors. It is a valid, reliable and sensitive instrument.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3543238/

Record 1390

pmcid: PMC3666586

pmid: 22777393

doi: 10.1038/pcan.2012.26

title: Does variation in either age at start of therapy or duration of therapy make chemoprevention with finasteride cost-effective?

authors

- Stewart SB
- Scales CD Jr
- Moul JW
- Reed SD

journal: Prostate cancer and prostatic diseases

journalAbbr: Prostate Cancer Prostatic Dis

year: 2012

pubdate: 2012 Dec

volume: 15

issue: 4

pages: 380-5

abstract: BACKGROUND: Incremental cost-effectiveness ratios (ICERs) of finasteride for prostate cancer prevention are consistent with estimates beyond $100 000 per quality-adjusted life-year (QALY). The majority of these analyses are based on chemoprevention starting in men aged 50-55 years. We sought to evaluate the impact of varying both age at commencement of therapy and length of therapy on the cost-effectiveness of finasteride. METHODS: A probabilistic Markov model was designed to estimate lifetime prostate health-related costs and quality-adjusted survival for men receiving or not receiving chemoprevention with finasteride. ICERs across scenarios varying age at start of therapy and duration of chemoprevention were compared. RESULTS: The ICER for men starting chemoprevention at age 50 and continuing to age 75 was $88 800 per QALY when assuming finasteride causes a constant risk reduction across all tumor grades (base case 1) and $142 300 per QALY when assuming a differential treatment effect according to Gleason score (base case 2). When starting age is increased, the ICERs trend downward and nadir at 65 years to $64 700 per QALY (base case 1) and $118 600 per QALY (base case 2). Altering duration of therapy had minimal impact. Patient-level experiences with finasteride and BPH significantly influenced the cost-effectiveness of chemoprevention. CONCLUSIONS: Initiating chemoprevention at ages when prostate cancer incidence is higher improves its cost-effectiveness

profile. Only when assuming a constant risk reduction for all tumor grades, did finasteride fall below $100 000 per QALY, but this finding was not upheld when accounting for side effects associated with the drug.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3666586/

Record 1391

pmcid: PMC3448916

pmid: 22471974

doi: 10.1111/j.1476-5381.2012.01969.x

title: Effects of flavocoxid, a dual inhibitor of COX and 5-lipoxygenase enzymes, on benign prostatic hyperplasia.

authors

- Altavilla D
- Minutoli L
- Polito F
- Irrera N
- Arena S
- Magno C
- Rinaldi M
- Burnett BP
- Squadrito F
- Bitto A

journal: British journal of pharmacology

journalAbbr: Br J Pharmacol

year: 2012

pubdate: 2012 Sep

volume: 167

issue: 1

pages: 95-108

abstract: BACKGROUND AND PURPOSE: Inflammation plays a key role in the development of benign prostatic hyperplasia (BPH). Eicosanoids derived from the COX and 5-lipoxygenase (5-LOX) pathways are elevated in the enlarging prostate. Flavocoxid is a novel flavonoid-based 'dual inhibitor' of the COX and 5-LOX enzymes. This study evaluated the effects of flavocoxid in experimental BPH. EXPERIMENTAL APPROACH: Rats were treated daily with testosterone propionate (3 mg·kg(-1) s.c.) or its vehicle for 14 days to induce BPH. Animals receiving testosterone were randomized to receive vehicle (1 mL·kg(-1) , i.p.) or flavocoxid (20 mg·kg(-1) , i.p.) for 14 days. Histological changes, eicosanoid content and mRNA and protein levels for apoptosis-related proteins and growth factors were assayed in prostate tissue. The effects of flavocoxid were also tested on human prostate carcinoma PC3 cells. KEY RESULTS: Flavocoxid reduced prostate weight and hyperplasia, blunted inducible expression of COX-2 and 5-LOX as well as the increased production of PGE(2) and leukotriene B(4) (LTB(4)), enhanced pro-apoptotic Bax and caspase-9 and decreased the anti-apoptotic Bcl-2 mRNA. Flavocoxid also reduced EGF and VEGF expression. In PC3 cells, flavocoxid stimulated apoptosis and inhibited growth factor expression. Flavocoxid-mediated induction of apoptosis was inhibited by the pan-caspase inhibitor, Z-VAD-FMK, in PC3 cells, suggesting an essential role of caspases in flavocoxid-mediated apoptosis during prostatic growth. CONCLUSION AND IMPLICATIONS: Our results show that a 'dual inhibitor' of the COX and 5-LOX enzymes, such as flavocoxid, might represent a rational approach to reduce BPH through modulation of eicosanoid production and a caspase-induced apoptotic mechanism.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3448916/

Record 1392

pmcid: PMC3434076

pmid: 22866946

doi: 10.1186/1471-2334-12-179

title: Efficacy and safety of a dual boosted protease inhibitor-based regimen, atazanavir and fosamprenavir/ritonavir, against HIV: experience in a pediatric population.

authors

- Rusconi S
- Giacomet V
- Mameli C
- Viganò A
- Viganò O
- Adorni F
- Galli M
- Zuccotti GV

journal: BMC infectious diseases

journalAbbr: BMC Infect Dis

year: 2012

pubdate: 2012 Aug 6

volume: 12

issue:

pages: 179

abstract: BACKGROUND: Although dual-boosted protease inhibitors regimen is not recommended in children with HIV infection, such a strategy could be useful in subjects with a complex resistance profile. This study was aimed at assessing the long term efficacy and safety of a double-boosted protease inhibitor combination, fosamprenavir (fAVP) and atazanavir/ritonavir (ATV/r) in a cohort of HIV-infected children and adolescents who had failed with nucleoside reverse transcriptase inhibitors. METHODS: Seven vertically infected children and adolescents who had previously failed highly active antiretroviral therapy and were resistant to nucleoside reverse transcriptase inhibitors, received a dual protease inhibitor (PI) regimen including fAVP plus ATV/r for 42 months. The patients were assessed at baseline, every month for the first 24 weeks of therapy and every 3 months until month 32. Physical examination, CD4+ cell count, HIV-RNA viral load, lipid profile and hepatic function were assessed throughout the follow up. RESULTS: During the study no serious adverse events were reported. CD4 absolute number increased over-time in all subjects. At baseline the median HIV-RNA was 6562 cp/mL (ranging 1048 -102772 cp/mL) and rapidly decreased below the limit of detection (50 cp/mL) after 2 months of the new treatment and remained undetectable in all cases through the entire study period. At the beginning of the study all cases showed a normal lipid profile. During the study period, 4/7 subjects showed total cholesterol, low density lipoprotein and triglyceride levels >97th cent.le for the males and 94th cent.le for the females. HDL cholesterol showed protective values. Hepatic enzymes remained stable during the entire observation, whereas total bilirubin showed toxicity II/III grade in 6/7 subjects. No change in fat redistribution and insulin resistance was observed. CONCLUSION: Dual-boosted protease inhibitor therapy was virologically and immunologically effective and it could be considered as a possible alternative to a rescue regimen in children and adolescents. However, hypercholesterolemia and hypertriglyceridemia need close follow-up and may limit the use of this therapeutic option.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3434076/

Record 1393

pmcid: PMC3755848

pmid: 24009916

doi:

title: Endocrine and metabolic disorders in β-thalassemiamajor patients.

authors

- Saffari F
- Mahyar A
- Jalilolgadr S

journal: Caspian journal of internal medicine

journalAbbr: Caspian J Intern Med

year: 2012

pubdate: 2012 Summer

volume: 3

issue: 3

pages: 466-72

abstract: BACKGROUND: Thalassemia is the most common hereditary anemia and beta thalassemia major is its most severe form. Endocrine abnormalities in thalassemia major are common disturbing complications that need prompt management. The purpose of this study was to determine the endocrine disorders and bone mineral density in patients with major -thalassemia in . METHODS: In this cross- sectional study, 77 patients with - thalassemia major (15-36 years old) were enrolled. Physical examination, laboratory tests, bone radiography and bone density measurements were performed. Then, the data were analyzed. RESULTS: Forty patients were males. The mean age was 21.26±4.53 years old. The mean BMI was 20.15±2.79 kg/m(2). Impaired puberty, short stature, hypothyroidism, diabetes mellitus, IGT, hypoparathyroidism, vitamin D deficiency and vitamin D insufficiency were observed in 46.8%, 33.8%, 18.2%, 16.9%, 13%, 7.8%, 45.5% and 24.7% of patients, respectively. Nearly 80% of patients had low bone mineral density. Bone mineral density was significantly associated with hypogonadism (p=0.001), short stature (p=0.026), hypoparathyroidism (p=0.031), hypothyroidism (p=0.048), diabetes mellitus (p=0.002) and vitamin D deficiency (p<0.001). CONCLUSION: Impaired puberty and short stature were the most common endocrine complications in our population. Low bone density (osteopenia, osteoporosis) is significantly different in β-thalassemic patients with and without endocrine complications.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3755848/

Record 1394

pmcid: PMC3441136

pmid: 23024707

doi: 10.1177/1756287212457115

title: Experience with the combination of dutasteride and tamsulosin in the long-term management of benign prostatic hyperplasia.

authors

- Chughtai B
- Elterman DS
- Lee R
- Te AE
- Kaplan SA

journal: Therapeutic advances in urology

journalAbbr: Ther Adv Urol

year: 2012

pubdate: 2012 Oct

volume: 4

issue: 5

pages: 267-72

abstract: Lower urinary tract symptoms (LUTS) caused by benign prostatic hyperplasia (BPH) commonly affect older men. These bothersome symptoms can lead to a decreased quality of life. Currently, two classes of drugs - α-adrenergic blockers and 5α-reductase inhibitors - are prescribed to treat LUTS secondary to BPH. Due to their different mechanisms of action, these medications work in a synergistic manner. Trials of combination therapy have been conducted to assess its effect compared with monotherapy. Current data support combination therapy in men with moderately enlarged prostates and moderate to severe symptoms.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3441136/

Record 1395

pmcid: PMC3271294

pmid: 22323868

doi: 10.3346/jkms.2012.27.2.194

title: Factors to predict positive results of gonadotropin releasing hormone stimulation test in girls with suspected precocious puberty.

authors

- Nam HK
- Rhie YJ
- Son CS
- Park SH
- Lee KH

journal: Journal of Korean medical science

journalAbbr: J Korean Med Sci

year: 2012

pubdate: 2012 Feb

volume: 27

issue: 2

pages: 194-9

abstract: Sometimes, the clinical findings and the results of the gonadotropin-releasing hormone (GnRH) stimulation test are inconsistent in girls with early breast development and bone age advancement. We aimed to investigate the factors predicting positive results of the GnRH stimulation test in girls with suspected central precocious puberty (CPP). We reviewed the records of 574 girls who developed breast budding before the age of 8 yr and underwent the GnRH stimulation test under the age of 9 yr. Positive results of the GnRH stimulated peak luteinizing hormone (LH) level were defined as 5 IU/L and over. Girls with the initial positive results (n = 375) showed accelerated growth, advanced bone age and higher serum basal LH, follicle-stimulating hormone, and estradiol levels, compared to those with the initial negative results (n = 199). Girls with the follow-up positive results (n = 64) showed accelerated growth and advanced bone age, compared to those with the follow-up negative results. In the binary logistic regression, the growth velocity ratio was the most significant predictive factor of positive results. We suggest that the rapid growth velocity is the most useful predictive factor for positive results in the GnRH stimulation test in girls with suspected precocious puberty.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3271294/

Record 1396

pmcid: PMC6493589

pmid: 23121864

doi: 10.1111/cns.12022

title: Goserelin cannot improve the weakness of a patient with kennedy disease after 40-week administration.

authors

- Shi ZG
- Jiang DK
- Zhao M
- Zhang SQ

journal: CNS neuroscience & therapeutics

journalAbbr: CNS Neurosci Ther

year: 2012

pubdate: 2012 Dec

volume: 18

issue: 12

pages: 1021-3

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6493589/

Record 1397

pmcid: PMC3420427

pmid: 22937319

doi: 10.1155/2012/194312

title: Granulocytic sarcoma of the male breast in acute myeloblastic leukemia with concurrent deletion of 5q and trisomy 8.

authors

- Rizwan M
- Islam MM
- Rehman ZU

journal: Case reports in hematology

journalAbbr: Case Rep Hematol

year: 2012

pubdate: 2012

volume: 2012

issue:

pages: 194312

abstract: We describe a unique case of Granulocytic Sarcoma (GS) in a male, who presented to us with a painless right breast mass without any prior history of Leukemia. GS is an extramedullary tumor of

myeloproliferative precursors and may involve multiple sites of the body, but involvement of male breast is extremely rare. In the absence of clinical history or hematological abnormality, GS may be misdiagnosed, depending on the degree of myeloid differentiation present within the tumor. Often it is misdiagnosed as lymphoma. Diagnosis is made by finding eosinophilic myelocytes, myeloperoxidase, chloroacetate esterase staining, and lysozyme immunostain. Chemotherapy regimens similar to acute myeloid leukemia are recommended to treat GS. Recognition of this rare entity is important because early, aggressive chemotherapy can induce regression of the tumor and improve patient longevity.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3420427/

Record 1398

pmcid: PMC3468794

pmid: 23091553

doi: 10.4048/jbc.2012.15.3.371

title: Granulomatous Mastitis during Chronic Antidepressant Therapy: Is It Possible a Conservative Therapeutic Approach?

authors

- Bellavia M
- Damiano G
- Palumbo VD
- Spinelli G
- Tomasello G
- Marrazzo A
- Ficarella S
- Bruno A
- Sammartano A
- Fiorentini T
- Scio A
- Maione C
- Lo Monte AI

journal: Journal of breast cancer

journalAbbr: J Breast Cancer

year: 2012

pubdate: 2012 Sep

volume: 15

issue: 3

pages: 371-2

abstract: Granulomatous mastitis is a rare benign inflammatory disease of the breast with multiple etiologies such as tuberculosis, sarcoidosis, foreign body reaction, and mycotic and parasitic infections. In contrast, idiopathic granulomatous mastitis (IGM) is characterized by the presence of chronic granulomatous lobulitis in the absence of an obvious etiology. Clinically and radiologically it may mimic breast carcinoma and so awareness of surgeons, pathologists, and radiologists is essential to avoid unnecessary mastectomies. Cases of IGM are reported during antidepressant therapy in patients also showing high levels of prolactinemia. In these cases, we believe that surgical excision must be avoided being replaced with a conservative management of the pathological condition based on a corticosteroid treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3468794/

Record 1399

pmcid: PMC3603073

pmid: 23565425

doi: 10.4103/2230-8210.104089

title: Gynaecomastia as the initial presentation of thyrotoxicosis.

authors

- Sanyal T
- Dutta D
- Shivprasad K
- Ghosh S
- Mukhopadhyay S
- Chowdhury S

journal: Indian journal of endocrinology and metabolism

journalAbbr: Indian J Endocrinol Metab

year: 2012

pubdate: 2012 Dec

volume: 16

issue: Suppl 2

pages: S352-3

abstract: Gynaecomastia is a well-recognized manifestation of thyrotoxicosis occurring in a widely variable percentage. Some studies have reported the percentage to be high as 40% while others report it being around 10%. The variation can be attributed to the difference in the criteria used to define gynae-comaslia or racial factors. However, thyrotoxicosis presenting with gynaceomastia as an intial manifestation is very rare. Only handful case reports are found in literature. Here we present a case of 35-year-old Indian male, with thyrotoxcosis who presented initially with painful bilateral gynaceomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3603073/

Record 1400

pmcid: PMC10876201

pmid: 22790552

doi: 10.1590/s1516-31802012000300009

title: Gynecomastia: physiopathology, evaluation and treatment.

authors

- Barros AC
- Sampaio Mde C

journal: Sao Paulo medical journal = Revista paulista de medicina

journalAbbr: Sao Paulo Med J

year: 2012

pubdate: 2012

volume: 130

issue: 3

pages: 187-97

abstract: Gynecomastia (GM) is characterized by enlargement of the male breast, caused by glandular proliferation and fat deposition. GM is common and occurs in adolescents, adults and in old age. The aim of this review is to discuss the pathophysiology, etiology, evaluation and therapy of GM. A hormonal imbalance between estrogens and androgens is the key hallmark of GM generation. The etiology of GM is attributable to physiological factors, endocrine tumors or dysfunctions, non-endocrine diseases, drug use or idiopathic causes. Clinical evaluation must address diagnostic confirmation, search for an etiological factor and classify GM into severity grades to guide the treatment. A proposal for tailored therapy is presented. Weight loss, reassurance, pharmacotherapy with tamoxifen and surgical correction are the therapeutic options. For long-standing GM, the best results are generally achieved through surgery, combining liposuction and mammary adenectomy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC10876201/

Record 1401

pmcid: PMC3662144

pmid: 23723555

doi: 10.4103/0253-7176.108233

title: Iloperidone-induced Galactorrhea in a Middle-aged Female.

authors

- Dutta A
- Barua S
- Dan A
- Chakraborty K
- Mandal M

journal: Indian journal of psychological medicine

journalAbbr: Indian J Psychol Med

year: 2012

pubdate: 2012 Oct

volume: 34

issue: 4

pages: 396-8

abstract: Iloperidone, a piperidinyl-benzisoxazole derivative, is structurally related to risperidone and approved for treatment in acute stage of schizophrenia. Iloperidone is usually considered as a prolactin sparing atypical antipsychotic thereby offering treatment advantage. We aim to present the first reported case of iloperidone-induced hyperprolactinemic galactorrhea in a middle-aged female. A middle-aged female with the diagnosis of paranoid schizophrenia was treated with iloperidone up to a dosage of 8 mg/day. Three months after starting the medicine, patient developed galactorrhea for which no other medical cause could be ascertained except for increased prolactin level. Iloperidone was stopped and aripiprazole was initiated with which galactorrhea subsided and prolactin level returned to normal. Index case report amply demonstrates that Iloperidone can cause hyperprolactinemic galactorrhea even at low dosage and after considerable period into the treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3662144/

Record 1402

pmcid: PMC3399643

pmid: 22584436

doi: 10.1634/theoncologist.2012-0039

title: Inflammatory breast cancer: what we know and what we need to learn.

authors

- Yamauchi H
- Woodward WA
- Valero V
- Alvarez RH
- Lucci A
- Buchholz TA
- Iwamoto T
- Krishnamurthy S
- Yang W
- Reuben JM
- Hortobágyi GN
- Ueno NT

journal: The oncologist

journalAbbr: Oncologist

year: 2012

pubdate: 2012

volume: 17

issue: 7

pages: 891-9

abstract: PURPOSE: We review the current status of multidisciplinary care for patients with inflammatory breast cancer (IBC) and discuss what further research is needed to advance the care of patients with this disease. DESIGN: We performed a comprehensive review of the English-language literature on IBC through computerized literature searches. RESULTS: Significant advances in imaging, including digital mammography, high-resolution ultrasonography with Doppler capabilities, magnetic resonance imaging, and positron emission tomography-computed tomography, have improved the diagnosis and staging of IBC. There are currently no established molecular criteria for distinguishing IBC from noninflammatory breast cancer. Such criteria would be helpful for the diagnosis and development of novel targeted therapies. Combinations of neoadjuvant systemic chemotherapy, surgery, and radiation therapy have led to an improved prognosis; however, the overall 5-year survival rate for patients with IBC remains very low (~30%). Sentinel lymph node biopsy and skin-sparing mastectomy are not recommended for patients with IBC. CONCLUSION: Optimal management of IBC requires close coordination among medical, surgical, and radiation oncologists, as well as radiologists and pathologists. There is a need to identify molecular changes that define the pathogenesis of IBC to enable eradication of IBC with the use of IBC-specific targeted therapies.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3399643/

Record 1403

pmcid: PMC3628259

pmid: 23599596

doi: 10.4103/0972-3919.108843

title: Initial evaluation of inflammatory breast cancer with fluorodeoxyglucose positron emission tomography.

authors

- Sager S

- Asa S
- Doner RK
- Leblebici C
- Halac M

journal: Indian journal of nuclear medicine : IJNM : the official journal of the Society of Nuclear Medicine, India

journalAbbr: Indian J Nucl Med

year: 2012

pubdate: 2012 Jan

volume: 27

issue: 1

pages: 33-4

abstract: Inflammatory breast cancer (IBC) is the most aggressive form of locally advanced breast cancer. We present here (18)F FDG PET/CT findings of two patients with IBC. These patients were referred to the Nuclear Medicine department for staging of IBC. FDG PET/CT images showed diffuse infiltration of breasts with multiple lymph nodes and multiple metastases in whole-body PET/CT images. FDG PET provides additional information regarding lymph nodes or distant metastases in the initial evaluation of IBC.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3628259/

Record 1404

pmcid: PMC3486776

pmid: 23125888

doi:

title: Invasive ductal breast carcinoma underneath a lipoma in a male patient.

authors

- Landero J
- Touloei K
- Glick BP

journal: The Journal of clinical and aesthetic dermatology

journalAbbr: J Clin Aesthet Dermatol

year: 2012

pubdate: 2012 Oct

volume: 5

issue: 10

pages: 33-7

abstract: Male breast cancer is a rare malignancy and accounts for less than one percent of all cancers in men. The authors describe the case of a 76-year-old Caucasian man with invasive ductal breast carcinoma who presented with a common lipoma. This paper reviews the current literature on epidemiology, risk factors, etiology, different types of breast cancer, clinical presentation, imaging, diagnostic workup, and treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3486776/

Record 1405

pmcid: PMC3485864

pmid: 23133775

doi: 10.1155/2012/352751

title: Male Child with Van Wyk-Grumbach's Syndrome and Other Complications of Long-Standing Primary Hypothyroidism: A Case Report.

authors

- Omran A
- Peng J
- Shrestha B
- Ashhab MU
- Yin F

journal: Case reports in pediatrics

journalAbbr: Case Rep Pediatr

year: 2012

pubdate: 2012

volume: 2012

issue:

pages: 352751

abstract: Primary hypothyroidism in the juvenile population generally leads to retardation of linear growth and delay or even arrest of puberty. However, in rare conditions, children with long-standing hypothyroidism present with signs of Van Wyk-Grumbach's syndrome (VWGS) which include juvenile hypothyroidism, delayed bone age, and pseudoprecocious puberty. We report a rare case of prepubertal male child from Asian origin, presented with long-standing untreated hypothyroidism complicated with VWGS and other complications including obesity, short stature, hepatomegaly, asymptomatic mild pericardial effusion, and pituitary hyperplasia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3485864/

Record 1406

pmcid: PMC3510963

pmid: 23226638

doi: 10.4103/2230-8210.102994

title: Male genitoplasty for 46 XX congenital adrenal hyperplasia patients presenting late and reared as males.

authors

- Sharma S
- Gupta DK

journal: Indian journal of endocrinology and metabolism

journalAbbr: Indian J Endocrinol Metab

year: 2012

pubdate: 2012 Nov

volume: 16

issue: 6

pages: 935-8

abstract: AIM: To evaluate the clinical profile and management of 46 XX Congenital Adrenal Hyperplasia (CAH) patients presenting with severe virilization and assigned a male gender. MATERIALS AND METHODS: Of 173 children diagnosed with CAH at the Pediatric Intersex Clinic since 1980, seven children with CAH presented late

with severe virilization and were reared as males. All of them were assigned the male sex with removal of the female adnexa. Six were treated with male genitoplasty. Appropriate hormonal supplementation was offered after puberty. RESULTS: The mean age at presentation was 14.2 years (7 - 21). Six patients had presented after puberty, only one at seven years of age. Staged male genitoplasty comprising of chordee correction, male urethroplasty, and bilateral testicular prosthesis was performed. The female adnexa (uterus, ovaries, most of the upper vagina, and the fallopian tubes) were removed. The mental makeup was masculine in six and bigender in one. Bilateral mastectomy was performed at puberty in all. Hormonal treatment comprised of glucocorticoids and testosterone. Six patients were comfortable with the outcome of the masculinizing genitoplasty. One had a short-sized phallus. One had repeated attacks of urinary tract infection arising from the retained lower vaginal pouch. Social adjustments were good in all, except in one who had a bigender mental makeup. CONCLUSION: CAH patients with severe virilization presenting late and reared as males are extremely rare. However, the assigned gender can be retained adequately as males, meeting the socioeconomic compulsions of the society. The results are satisfactory following appropriate surgical procedures and hormonal supplementation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3510963/

Record 1407

pmcid: PMC3507397

pmid: 23204656

doi: 10.4103/0970-1591.102703

title: Managing disorder of sexual development surgically: A single center experience.

authors

- Kumar J
- Kumar V
- Bhatia V
- Dabadghao P
- Chaturvedi S
- Kapoor R
- Ansari MS

journal: Indian journal of urology : IJU : journal of the Urological Society of India

journalAbbr: Indian J Urol

year: 2012

pubdate: 2012 Jul

volume: 28

issue: 3

pages: 286-91

abstract: INTRODUCTION: Ambiguous genitalia are a major cause of parental anxiety and create psychological and social problems to patient, if not managed properly. Here we present our experience in managing patients with ambiguous genitalia. MATERIAL AND METHODS: We retrospectively reviewed clinical records of all patients with ambiguous genitalia managed surgically at out institute between December 1989 and January 2011. Relevant history, clinical examination, investigations and surgical procedures performed were analyzed and results were evaluated in terms of anatomical, functional and psychosexual outcomes. RESULTS: Female pseudohermaphroditism was the most common cause of genital ambiguity in our patients. Male and female genitoplasty was done according to gender of rearing, genital anatomy and parental choice. Twenty six patients (86.6%) reported satisfactory cosmetic outcome and 22 (73.3%) satisfactory functional outcome on long term follow-up. Among the 24 patients diagnosed as male pseudohermaphroditism 14 (82.3%) patient have reported satisfactory cosmetic outcome and 13 (76.4%) have reported satisfactory functional outcome. In patients with mixed gonadal dysgenesis and true hermaphroditism satisfactory cosmetic and functional outcome was seen in

70% patient. CONCLUSION: Managing patients of genital ambiguity according to gender of rearing, genital anatomy and parental choice carries good prognosis in terms of anatomical, functional and psychosexual outcome.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3507397/

Record 1408

pmcid: PMC3359955

pmid: 22640971

doi: 10.1186/1750-1172-7-S1-S4

title: McCune-Albright syndrome and the extraskeletal manifestations of fibrous dysplasia.

authors

- Collins MT
- Singer FR
- Eugster E

journal: Orphanet journal of rare diseases

journalAbbr: Orphanet J Rare Dis

year: 2012

pubdate: 2012 May 24

volume: 7 Suppl 1

issue: Suppl 1

pages: S4

abstract: Fibrous dysplasia (FD) is sometimes accompanied by extraskeletal manifestations that can include any combination of café-au-lait macules, hyperfunctioning endocrinopathies, such as gonadotropin-independent precocious puberty, hyperthyroidism, growth hormone excess, FGF23-mediated renal phosphate wasting, and/or Cushing syndrome, as well as other less common features. The combination of any of these findings, with or without FD, is known as McCune-Albright syndrome (MAS). The broad spectrum of involved tissues and the unpredictable combination of findings owe to the fact that molecular defect is due to dominant activating mutations in the widely expressed signaling protein, Gsα, and the fact these mutations arises sporadically, often times early in development, prior to gastrulation, and can distribute across many or few tissues.The complexity can be mastered by a systematic screening of potentially involved tissues and cognizance that the pattern of involved tissues is established, to some degree, in utero. Thorough testing allows the clinician to establish, often times at presentation, the full extent of the disease, and importantly as well what tissues are unaffected. Treatment and follow-up can then be focused on affected systems and a meaningful prognosis can be offered to the patient and family. The authors outline screening and treatment strategies that allow for effective management of the extraskeletal manifestations of FD.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3359955/

Record 1409

pmcid: PMC3513934

pmid: 23227100

doi: 10.4103/1742-6413.102863

title: Metastatic hepatocellular carcinoma presenting as gynecomastia in male: A diagnostic dilema in fine needle aspiration cytology.

authors

- Nasit JG

- Shah B
- Shah M

journal: CytoJournal

journalAbbr: Cytojournal

year: 2012

pubdate: 2012

volume: 9

issue:

pages: 21

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3513934/

Record 1410

pmcid: PMC3425294

pmid: 22923833

doi: 10.4103/0019-5049.98782

title: Nerve stimulator-guided thoracic paravertebral block for gynecomastia surgery.

authors

- Jadon A

journal: Indian journal of anaesthesia

journalAbbr: Indian J Anaesth

year: 2012

pubdate: 2012 May

volume: 56

issue: 3

pages: 298-300

abstract: Thoracic paravertebral block (TPVB) is gaining popularity for female breast surgeries due to various advantages like less nausea and vomiting and better post-operative pain relief, which helps in early ambulation and discharge from the hospital. Use of nerve stimulator during this block has further enhanced its success and safety profile. Male breast surgery is usually done either under general anaesthesia or local infiltrative anaesthesia combining with intravenous sedation. We postulated that the advantages of TPVB could be helpful for early mobilization and discharge of minor breast surgery in male patients. However, to our knowledge, there is no such report suggestive of TPVB for exclusive male breast surgery. We used nerve stimulator-guided TPVB for gynecomastia surgery in two patients where general anaesthesia was not feasible. Both patients had successful block and showed good post-operative recovery and were discharged on the same day. They had long post-operative pain relief without any block-related complication. A case report of two such cases of gynecomastia surgery (male breast surgery) done under TPVB is presented.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3425294/

Record 1411

pmcid: PMC3397185

pmid: 23730053

doi: 10.1007/s12262-012-0590-9

title: Oncoplastic breast surgery.

authors

- Pillarisetti RR
- Querci Della Rovere G

journal: The Indian journal of surgery

journalAbbr: Indian J Surg

year: 2012

pubdate: 2012 Jun

volume: 74

issue: 3

pages: 255-63

abstract: Breast Surgery is now a recognized subspecialty of General Surgery abroad with structured training for designated 'Oncoplastic Breast Surgeons'. Oncoplastic Breast surgery is probably one of the most interesting and challenging new developments over the past 20 years. The aims of Oncoplastic surgery are wide local excision of the cancer coupled with partial reconstruction of the defect to achieve a cosmetically acceptable result. Avoidance of mastectomy and consequent reduction of psychological morbidity are the principal goals in the development of various oncoplastic techniques. The use of plastic surgical techniques not only ensures good cosmetic outcome, but also allows the cancer surgeon to remove the tumour with greater volume of surrounding tissue, thus extending the boundaries of breast conserving surgery. Proper patient selection and careful planning after proper radiological and clinical assessment are the two essential prerequisites before undertaking oncoplastic breast surgery. Oncoplastic surgery involves both volume displacement and volume replacement techniques. Some commonly used volume displacement procedures are described in the article. The need for adjustment of contralateral breast should also be anticipated at the time of planning breast conserving surgery, which can be done either at the same time as breast cancer surgery or as a delayed setting.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3397185/

Record 1412

pmcid: PMC3354934

pmid: 22701839

doi: 10.4103/2230-8210.94250

title: Pathogenesis and treatment of human immunodeficiency virus lipodystrophy.

authors

- Jain SS
- Ramteke KB
- Raparti GT
- Kalra S

journal: Indian journal of endocrinology and metabolism

journalAbbr: Indian J Endocrinol Metab

year: 2012

pubdate: 2012 Mar

volume: 16 Suppl 1

issue: Suppl1

pages: S20-6

abstract: Enhanced understanding about the way human immunodeficiency virus (HIV) infects and causes infection in humans has led to invention and use of newer more effective antiretroviral drugs. As treatment for HIV is long term, side effects of the antiretrovirals become an important area of research focus. Antiretrovirals can cause severe metabolic abnormalities, collectively known as HIV lipodystrophy syndrome. If untreated, these metabolic abnormalities have the potential to increase stroke and cardiac ischemia. Management includes choice of nonoffending drugs, switch over to less toxic drugs, hypolipidemics, oral antidiabetics including thiazolidinediones, metformin and growth hormone analogs and finally facial surgeries. Updated knowledge about HIV lipodystrophy, and the hormone-related drugs used to treat it, is essential for physicians and endocrinologists to be able to diagnose the patients and effectively treat them.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3354934/

Record 1413

pmcid: PMC3386774

pmid: 22672861

doi: 10.4274/jcrpe.615

title: Plasma kisspeptin levels in girls with premature thelarche.

authors

- Akinci A
- Cetin D
- Ilhan N

journal: Journal of clinical research in pediatric endocrinology

journalAbbr: J Clin Res Pediatr Endocrinol

year: 2012

pubdate: 2012 Jun

volume: 4

issue: 2

pages: 61-5

abstract: OBJECTIVE: Premature thelarche (PT) is defined as isolated breast development without secondary sex characteristics in girls below the age of eight. We aimed to determine whether the level of kisspeptin, which plays a role in the release of gonadotropins, is associated with PT. METHODS: The patient group included children with PT aged 3-8 years (n=20) and the control group included healthy children in the same age range (n=20). Height standard deviation scores (HSDSs), bone maturation and growth velocity were evaluated in the two groups. Basal follicle-stimulating hormone (FSH), luteinizing hormone (LH), estradiol (E2), prolactin (PRL), and sex hormone-binding globulin (SHBG) levels were also measured in the two groups by immunochemiluminometric assay (ICMA). A gonadotropin-releasing hormone (GnRH) test was also conducted in the patient group and the peak levels of FSH and LH were determined. Kisspeptin levels were measured using enzyme immunoassay (EIA). RESULTS: No differences were found between the groups in terms of age, HSDS, annual growth rate and bone age. While the plasma basal FSH, LH and E2 levels in the patient and control groups did not show statistically significant differences, PRL levels were higher in the patient group ($p<0.05$). Peak LH response to GnRH test was at the prepubertal level (<5 ng/mL) in patients with PT. In the patient group, kisspeptin levels were significantly higher compared to the levels in the control group (2.96 ± 1.21 ng/dL vs. 1.19 ± 0.41 ng/dL, $p<0.05$), and kisspeptin levels showed a significant correlation with PRL, FSH, LH, and E2 levels ($p<0.05$). CONCLUSIONS: In this study, plasma kisspeptin levels were found to be higher in patients with PT and to show a positive correlation with increased PRL levels. Kisspeptin is one of the neuropeptides that plays a role in the onset of puberty. Our results support the hypothesis that PT may result from the temporary activation of central stimulants.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3386774/

Record 1414

pmcid: PMC3400165

pmid: 22892919

doi: 10.6061/clinics/2012(07)09

title: Postural control in women with breast hypertrophy.

authors

- Barbosa AF
- Raggi GC
- Sá Cdos S
- Costa MP
- Lima JE Jr
- Tanaka C

journal: Clinics (Sao Paulo, Brazil)

journalAbbr: Clinics (Sao Paulo)

year: 2012

pubdate: 2012 Jul

volume: 67

issue: 7

pages: 757-60

abstract: OBJECTIVES: The consequences of breast hypertrophy have been described based on the alteration of body mass distribution, leading to an impact on psychological and physical aspects. The principles of motor control suggest that breast hypertrophy can lead to sensorimotor alterations and the impairment of body balance due to postural misalignment. The aim of this study is to evaluate the postural control of women with breast hypertrophy under different sensory information conditions. METHOD: This cross-sectional study included 14 women with breast hypertrophy and 14 without breast hypertrophy, and the mean ages of the groups were 39 ± 15 years and 39 ± 16 years, respectively. A force platform was used to assess the sensory systems that contribute to postural control: somatosensory, visual and vestibular. Four postural conditions were sequentially tested: eyes open and fixed platform, eyes closed and fixed platform, eyes open and mobile platform, and eyes closed and mobile platform. The data were processed, and variables related to the center of pressure were analyzed for each condition. The Kruskal-Wallis test was used to compare the conditions between the groups for the area of center of pressure displacement and the velocity of center of pressure displacement in the anterior-posterior and medial-lateral directions. The alpha level error was set at 0.05. RESULTS: Women with breast hypertrophy presented an area that was significantly higher for three out of four conditions and a higher velocity of center of pressure displacement in the anterior-posterior direction under two conditions: eyes open and mobile platform and eyes closed and mobile platform. CONCLUSIONS: Women with breast hypertrophy have altered postural control, which was demonstrated by the higher area and velocity of center of pressure displacement.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3400165/

Record 1415

pmcid: PMC3432225

pmid: 22970062

doi: 10.2349/biij.8.1.e6

title: Precocious puberty in children: A review of imaging findings.

authors

- Faizah M
- Zuhanis A
- Rahmah R
- Raja A
- Wu L
- Dayang A
- Zulfiqar M

journal: Biomedical imaging and intervention journal

journalAbbr: Biomed Imaging Interv J

year: 2012

pubdate: 2012 Jan

volume: 8

issue: 1

pages: e6

abstract: OBJECTIVES: This review was aimed at determining the imaging findings in patients with precocious puberty. RESULTS: Within a period of 8 years (from 2002 to 2010) there were 53 patients diagnosed with precocious puberty. Out of the 53 patients, 37 had undergone diagnostic imaging to detect the possible organic causes of precocious puberty. Imaging findings were positive in 31 patients and out of that, 3 patients had 2 findings each (34 abnormalities). Of the patients with positive imaging findings, central precocious puberty (gonadotrophin-dependent) was more common (81%; 25/31) and the causes included: tuber cinereum hamartoma (n = 10), glioma (n = 6), pineal gland tumour (n = 4), hydrocephalous (n = 3), arachnoid cyst (n = 2) and others (n = 3). Peripheral precocious puberty (gonadotrophin-independent) causes included: testicular adrenal rest tumour (n = 3), adrenal carcinoma (n = 1), ovarian granulosa thecal cell tumour (n = 1), and tuberous sclerosis (n = 1). CONCLUSION: Positive imaging findings were observed in 84% (31/37) of the subjects. Hypothalamic hamartoma was the most common imaging finding in central precocious puberty while testicular adrenal rest tumour was the most common imaging finding in peripheral precocious puberty.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3432225/

Record 1416

pmcid: PMC3603023

pmid: 23565375

doi: 10.4103/2230-8210.104036

title: Precocious puberty in girls.

authors

- Chittwar S
- Shivprakash
- Ammini AC

journal: Indian journal of endocrinology and metabolism

journalAbbr: Indian J Endocrinol Metab

year: 2012

pubdate: 2012 Dec

volume: 16

issue: Suppl 2

pages: S188-91

abstract: Precocious puberty in girls can be due to number of factors of which idiopathic central precocious puberty is the most common etiology. Here, we describe 3 cases of precocious puberty where the first case had premature thelarche in the background history of mother with Type 2 Diabetes Mellitus, cases 2 and 3 had ovarian tumours with heterogeneity in presentation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3603023/

Record 1417

pmcid: PMC3414798

pmid: 22734844

doi: 10.1186/1752-1947-6-162

title: Precocious puberty secondary to a mixed germ cell-sex cord-stromal tumor associated with an ovarian yolk sac tumor: a case report.

authors

- Metwalley KA
- Elsers DA
- Farghaly HS
- Abdel-Lateif H
- Abdel-Kader M

journal: Journal of medical case reports

journalAbbr: J Med Case Rep

year: 2012

pubdate: 2012 Jun 26

volume: 6

issue:

pages: 162

abstract: INTRODUCTION: Ovarian tumors are the least common cause of sexual precocity in girls. Mixed germ cell-sex cord-stromal tumors associated with a yolk sac tumor of the ovary are rare neoplasms, of which only a small number of well-documented cases have been described so far. Here, we report precocious puberty in a four-year-old Egyptian girl caused by a mixed germ cell-sex cord-stromal tumor associated with a yolk sac tumor of the ovary. CASE PRESENTATION: A four-year-old Egyptian girl was referred to our pediatric endocrinology unit for evaluation of bilateral breast budding, pubic hair and vaginal bleeding. On examination, we found that her breast enlargement and pubic hair were compatible with Tanner III. A thorough workup revealed a large mass in her right ovary. Magnetic resonance imaging ofher brain showed that her pituitary gland was normal. A hormonal assay revealed high levels of estradiol, 280 to 375pmol/L; progesterone, 5.3 nmol/L; testosterone 38.9 pg/mL; and androstenedione, 4.1 ng/mL. Her basal and stimulated levels of luteinizing hormone and follicle-stimulating hormone were low. Tumor markers levels were high, with a total inhibin of 1,069U/L and an alpha-fetoprotein of 987 μg/L. Her chromosomes were normal (46XX). Our patient underwent an explorative laparotomy and a solid tumor localized to her right ovary was identified. A right salpingo-oophorectomy was performed and the histopathological diagnosis was a mixed germ cell-sex cord-stromal tumorwith a yolk sac tumor of the ovary. Postoperatively, she was started on treatment with chemotherapy. Our patient is doing well without evidence of tumor recurrence or metastasis during eight months of postoperative follow-up. CONCLUSION: Although a mixed germ cell-sex cord-stromal tumor associated with a yolk sac tumor of the ovary is a rare occurrence, it should be considered in the differential diagnosis for a prepubescent girl with an abdominal mass and precocious puberty.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3414798/

Record 1418

pmcid: PMC3474114

pmid: 22665932

doi: 10.1259/bjr/78413721

title: Primary and secondary breast lymphoma: prevalence, clinical signs and radiological features.

authors

- Surov A
- Holzhausen HJ
- Wienke A
- Schmidt J
- Thomssen C
- Arnold D
- Ruschke K
- Spielmann RP

journal: The British journal of radiology

journalAbbr: Br J Radiol

year: 2012

pubdate: 2012 Jun

volume: 85

issue: 1014

pages: e195-205

abstract: OBJECTIVES: The purpose of this study was to determine the prevalence, clinical signs and radiological features of breast lymphoma. METHODS: This is a retrospective review of 36 patients with breast lymphoma (22 primary and 14 secondary). 35 patients were female and 1 was male; their median age was 65 years (range 24-88 years). In all patients, the diagnosis was confirmed histopathologically. RESULTS: The prevalence of breast lymphoma was 1.6% of all identified cases with non-Hodgkin lymphoma and 0.5% of cases with breast cancer. B-cell lymphoma was found in 94% and T-cell lymphoma in 6%. 96 lesions were identified (2.7 per patient). The mean size was 15.8 ± 8.3 mm. The number of intramammary lesions was higher in secondary than in primary lymphoma. The size of the identified intramammary lesions was larger in primary than in secondary lymphoma. Clinically, 86% of the patients presented with solitary or multiple breast lumps. In 14%, breast involvement was diagnosed incidentally during staging examinations. CONCLUSION: On mammography, intramammary masses were the most commonly seen (27 patients, 82%). Architectural distortion occurred in three patients (9%). In three patients (9%), no abnormalities were found on mammography. On ultrasound, the identified lesions were homogeneously hypoechoic or heterogeneously mixed hypo- to hyperechoic. On MRI, the morphology of the lesions was variable. After intravenous administration of contrast medium, a marked inhomogeneous contrast enhancement was seen in most cases. On CT, most lesions presented as circumscribed round or oval masses with moderate or high enhancement.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3474114/

Record 1419

pmcid: PMC3285737

pmid: 22396681

doi: 10.4306/pi.2012.9.1.25

title: Psychosocial aspects in girls with idiopathic precocious puberty.

authors

- Kim EY
- Lee MI

journal: Psychiatry investigation

journalAbbr: Psychiatry Investig

year: 2012

pubdate: 2012 Mar

volume: 9

issue: 1

pages: 25-8

abstract: OBJECTIVE: Precocious puberty leads to discrepancies between physical and chronological age. The aim of the present study was to evaluate behavioral patterns and social competences in 34 girls with idiopathic precocious puberty. METHODS: Height, body weight, and BMI were determined, and pubertal development was assessed. The Korean-child behavior checklist (K-CBCL) was completed by parents. RESULTS: 1) Body weight and BMI were significantly higher in precocious puberty group than in the control group. 2) T scores of total social competence and school scales of K-CBCL were significantly higher in the precocious puberty group than in the control group. 3) T scores of externalizing problems and total behavior problems of K-CBCL were also significantly higher in the precocious puberty group. 4) T scores of thought problems and attention problems of K-CBCL were significantly higher in the precocious puberty group than the control group. CONCLUSION: Although girls with precocious puberty had more behavioral problems, no significant differences were observed between the two study groups in terms of clinically-important T scores. However, the concerns of parents and patients regarding behavioral and emotional changes might be most important when evaluating psychosocial problems. Early recognition, sufficient understanding, communication with patients and parents regarding physical developmental difference from children in the same age, can improve the self-image of those with precocious puberty.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3285737/

Record 1420

pmcid: PMC3425456

pmid: 22943024

doi: 10.4088/PCC.11l01284

title: Quetiapine-induced galactorrhea with normal prolactin level in an adult female patient.

authors

- Mushtaq S
- Khan S
- Patel H

journal: The primary care companion for CNS disorders

journalAbbr: Prim Care Companion CNS Disord

year: 2012

pubdate: 2012

volume: 14

issue: 2

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3425456/

Record 1421

pmcid: PMC3459224

pmid: 23050167

doi: 10.5402/2012/934965

title: Surgery for Gynecomastia in the Islamic Golden Age: Al-Tasrif of Al-Zahrawi (936-1013 AD).

authors

- Chavoushi SH
- Ghabili K
- Kazemi A
- Aslanabadi A
- Babapour S
- Ahmedli R
- Golzari SE

journal: ISRN surgery

journalAbbr: ISRN Surg

year: 2012

pubdate: 2012

volume: 2012

issue:

pages: 934965

abstract: The rise of European science during the Renaissance is greatly indebted to the flourishing of the sciences during the Islamic Golden Age. However, some believe that medieval Islamic physicians and in particular surgeons had been merely a medium for Greco-Roman ideas. Contrarily, in some medieval Islamic medical books, such as Al-Tasrif of Al-Zahrawi (936-1013), the surgical instructions represent a change in the usual techniques or are accompanied by a case history, implying that the procedure was actually undertaken. Along with the hundreds of chapters on different diseases and related medical and surgical treatments, Al-Tasrif includes a chapter on surgical techniques for gynecomastia. The present paper is a review of the description of the surgical management of gynecomastia by Al-Zahrawi as well as that of the ancient Greek, medieval, and modern medicine. Although Al-Zahrawi seemed to base his descriptions of surgery for gynecomastia upon those of Paulus of Aegina, his modification of the procedure and application of the medicinal substances might be indicative of Al-Zahrawi's own practice of the procedure. Al-Zahrawi's surgical procedures remained unchanged for many centuries thenceforward until the technological evolution in the recent centuries.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3459224/

Record 1422

pmcid: PMC3464149

pmid: 22925442

doi: 10.1186/1741-7015-10-96

title: Tamoxifen for the management of breast events induced by non-steroidal antiandrogens in patients with prostate cancer: a systematic review.

authors

- Kunath F
- Keck B
- Antes G

- Wullich B
- Meerpohl JJ

journal: BMC medicine

journalAbbr: BMC Med

year: 2012

pubdate: 2012 Aug 28

volume: 10

issue:

pages: 96

abstract: BACKGROUND: Tamoxifen has emerged as a potential management option for gynecomastia and breast pain due to non-steroidal antiandrogens, and it is considered an alternative to surgery or radiotherapy. The objective of this systematic review was to assess the benefits and harms of tamoxifen, in comparison to other treatment options, for either the prophylaxis or treatment of breast events induced by non-steroidal antiandrogens in prostate cancer patients. METHODS: We searched CENTRAL, MEDLINE, EMBASE, reference lists, the abstracts of three major conferences and three trial registers to identify ongoing randomized controlled trials (RCTs). Two authors independently screened the articles identified, assessed the trial quality and extracted data. The protocol was prospectively registered (CRD42011001320; http://www.crd.york.ac.uk/PROSPERO). RESULTS: Four studies were identified. Tamoxifen significantly reduced the risk of suffering from gynecomastia (risk ratio 9RR0 0.10, 95% CI 0.05 to 0.22) or breast pain (RR 0.06, 95% CI 0.02 to 0.17) at six months compared to untreated controls. Tamoxifen also showed a significant benefit for the prevention of gynecomastia (RR 0.22, 95% CI 0.08 to 0.58) and breast pain (RR 0.25, 95% CI 0.10 to 0.64) when compared to anastrozole after a median of 12 months. One study showed a significant benefit of tamoxifen for the prevention of gynecomastia (RR 0.24, 95% CI 0.09 to 0.65) and breast pain (RR 0.20, 95% CI 0.06 to 0.65) when compared with radiotherapy at six months. Radiotherapy increased the risk of suffering from nipple erythema and skin irritation, but there were no significant differences for any other adverse events (all P>0.05). CONCLUSIONS: The currently available evidence suggests good efficacy of tamoxifen for the prevention and treatment of breast events induced by non-steroidal antiandrogens. The impact of tamoxifen therapy on long-term adverse events, disease progression and survival remains unclear. Further large, well-designed RCTs, including long-term follow-ups, are warranted. Also, the optimal dose needs to be clarified.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3464149/

Record 1423

pmcid: PMC3359659

pmid: 22654902

doi: 10.1155/2012/581712

title: The 2011 WPATH Standards of Care and Penile Reconstruction in Female-to-Male Transsexual Individuals.

authors

- Selvaggi G
- Dhejne C
- Landen M
- Elander A

journal: Advances in urology

journalAbbr: Adv Urol

year: 2012

pubdate: 2012

volume: 2012

issue:

pages: 581712

abstract: The World Professional Association for Transgender Health (WPATH) currently publishes the Standards of Care (SOC), to provide clinical guidelines for health care of transsexual, transgender and gender non-conforming persons in order to maximize health and well-being by revealing gender dysphoria. An updated version (7th version, 2011) of the WPATH SOC is currently available. Differences between the 6th and the 7th versions of the SOC are shown; the SOC relevant to penile reconstruction in female-to-male (FtM) persons are emphasized, and we analyze how the 2011 WPATH SOC is influencing the daily practice of physicians involved in performing a penile reconstruction procedure for these patients. Depending by an individual's goals and expectations, the most appropriate surgical technique should be performed: the clinic performing penile reconstruction should be able to offer the whole range of techniques, such as: metoidioplasty, pedicle and free flaps phalloplasty procedures. The goals that physicians and health care institutions should achieve in the next years, in order to improve the care of female-to-male persons, consist in: informing in details the individuals applying for penile reconstruction about all the implications; referring specific individuals to centers capable to deliver a particular surgical technique; implementing the surgery with the most updated refinements.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3359659/

Record 1424

pmcid: PMC3449135

pmid: 23008780

doi: 10.5402/2012/878635

title: The multifaceted granulosa cell tumours-myths and realities: a review.

authors

- Kanthan R
- Senger JL
- Kanthan S

journal: ISRN obstetrics and gynecology

journalAbbr: ISRN Obstet Gynecol

year: 2012

pubdate: 2012

volume: 2012

issue:

pages: 878635

abstract: Background. Granulosa cell tumors (GCTs), representing ~2% of ovarian tumours, are poorly understood neoplasms with unpredictable and undetermined biological behaviour. Design. 5 unusual presentations of GCT and a retrospective 14-year (1997-2011) surgical pathology review based on patient sex, age, tumour type and concurrent pathology findings are presented to discuss the "myths and realities" of GCTs in the context of relevant evidence-based literature. Results. The 5 index cases included (1) a 5 month-old boy with a left testicular mass, (2) a 7-day-old neonate with a large complex cystic mass in the abdomen, (3) a 76-year-old woman with an umbilical mass, (4) a 64-year-old woman with a complex solid-cystic pelvic mass, and (5) a 45 year-old woman with an acute abdomen. Pathological analysis confirmed the final diagnosis as (1) juvenile GCT, (2) macrofollicular GCT, (3) recurrent GCT 32 years later, (4) collision tumour: colonic adenocarcinoma and GCT, and (5) ruptured GCT. Conclusion. GCT is best considered as an unusual indolent neoplasm of low malignant potential with late recurrences that can arise in the ovaries and testicles in both the young and the old. Multifaceted clinical presentations coupled with the unpredictable biological behaviour with late relapses are diagnostic pitfalls necessitating a high degree of suspicion for accurate clinical and

pathological diagnosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3449135/

Record 1425

pmcid: PMC3235775

pmid: 22203832

doi: 10.1155/2012/103063

title: The pathophysiology of HIV-/HAART-related metabolic syndrome leading to cardiovascular disorders: the emerging role of adipokines.

authors

- Palios J
- Kadoglou NP
- Lampropoulos S

journal: Experimental diabetes research

journalAbbr: Exp Diabetes Res

year: 2012

pubdate: 2012

volume: 2012

issue:

pages: 103063

abstract: Individuals infected with human immunodeficiency virus (HIV) frequently demonstrate metabolic syndrome (MS) associated with increased incidence of cardiovascular disorders. Characteristics of HIV infection, such as immunodeficiency, viral load, and duration of the disease, in addition to the highly active antiretroviral therapy (HAART) have been suggested to induce MS in these patients. It is well documented that MS involves a number of traditional cardiovascular risk factors, like glucose, lipids, and arterial blood pressure abnormalities, leading to extensive atherogenic arterial wall changes. Nevertheless, the above traditional cardiovascular risk factors merely explain the exacerbated cardiovascular risk in MS. Nowadays, the adipose-tissue derivatives, known as adipokines, have been suggested to contribute to chronic inflammation and the MS-related cardiovascular disease. In view of a novel understanding on how adipokines affect the pathogenesis of HIV/HAART-related MS and cardiovascular complications, this paper focuses on the interaction of the metabolic pathways and the potential cardiovascular consequences. Based on the current literature, we suggest adipokines to have a role in the pathogenesis of the HIV/HAART-related MS. It is crucial to understand the pathophysiology of the HIV/HAART-related MS and apply therapeutic strategies in order to reduce cardiovascular risk in HIV patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3235775/

Record 1426

pmcid: PMC3518018

pmid: 23233900

doi: 10.5999/aps.2012.39.6.675

title: Unilateral gynecomastia in a tennis player.

authors

- Kang SG
- Song WJ
- Kim CH

- Kim JW
- Tark MS

journal: Archives of plastic surgery

journalAbbr: Arch Plast Surg

year: 2012

pubdate: 2012 Nov

volume: 39

issue: 6

pages: 675-8

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3518018/

Record 1427

pmcid: PMC3782199

pmid: 24175244

doi: 10.5527/wjn.v1.i3.69

title: Vascular access today.

authors

- Pantelias K
- Grapsa E

journal: World journal of nephrology

journalAbbr: World J Nephrol

year: 2012

pubdate: 2012 Jun 6

volume: 1

issue: 3

pages: 69-78

abstract: The number of patients with chronic kidney disease requiring renal replacement therapy has increased worldwide. The most common replacement therapy is hemodialysis (HD). Vascular access (VA) has a key role for successful treatment. Despite the advances that have taken place in the field of the HD procedure, few things have changed with regards to VA in recent years. Arteriovenous fistula (AVF), polytetrafluoroethylene graft and the cuffed double lumen silicone catheter are the most common used for VA. In the long term, a number of complications may present and more than one VA is needed during the HD life. The most common complications for all of VA types are thrombosis, bleeding and infection, the most common cause of morbidity in these patients. It has been estimated that VA dysfunction is responsible for 20% of all hospitalizations. The annual cost of placing and looking after dialysis VA in the United States exceeds 1 billion dollars per year. A good functional access is also vital in order to deliver adequate HD therapy. It seems that the native AVF that Brescia and Cimino described in 1966 still remains the first choice for VA. The native forearm AVFs have the longest survival and require the fewest interventions. For this reason, the forearm AVF is the first choice, followed by the upper-arm AVF, the arteriovenous graft and the cuffed central venous catheter is the final choice. In conclusion, VA remains the most important issue for patients on HD and despite the technical improvements, a number of problems and complications have to be resolved.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3782199/

Record 1428

pmcid: PMC3446187

pmid: 23056821

doi:

title: A Case of Atypical McCune-Albright Syndrome with Vaginal Bleeding.

authors

- Rostampour N
- Hashemipour M
- Kelishadi R
- Hovsepian S
- Hekmatnia A

journal: Iranian journal of pediatrics

journalAbbr: Iran J Pediatr

year: 2011

pubdate: 2011 Sep

volume: 21

issue: 3

pages: 399-403

abstract: BACKGROUND: McCune-Albright syndrome (MAS) is a rare non-inherited disorder characterized by the clinical triad of precocious puberty, cafe-au-lait skin lesions, and fibrous dysplasia of bone. CASE PRESENTATION: We report a girl with MAS, presenting initially with vaginal bleeding at the age of 17 months. Ultrasonography revealed unilateral ovarian cysts and ureteral and ovarian enlargement. Considering the clinical and paraclinical findings, the patient diagnosed as a case of gonadotropin-independent precocious puberty was treated with medroxy-progestrone acetate (MPA) for three months. During the follow up, recurrent episodes of bleeding, ovarian activation and cyst formation, as well as breast size development were reported. At the age of 5.5 years, fibrous dysplasia was detected, which in coexistence with precocious puberty confirmed the diagnosis of MAS. The patient had no cafe-au-lait skin macles during follow up. CONCLUSION: Considering that clinical manifestations of MAS appear later in the course of recurrent periods of ovarian activation and cyst formation, a careful clinical observation and follow up of patients is necessary and the diagnosis of MAS must be kept in mind in cases with gonadotropin-independent precocious puberty.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3446187/

Record 1429

pmcid: PMC3254848

pmid: 22247806

doi: 10.3802/jgo.2011.22.4.288

title: A mediastinal germ cell tumor mimicking an ectopic pregnancy.

authors

- Rivera C
- Poingt M
- Vandenbossche F
- Jougon J

journal: Journal of gynecologic oncology

journalAbbr: J Gynecol Oncol

year: 2011

pubdate: 2011 Dec

volume: 22

issue: 4

pages: 288-91

abstract: The objective is to report the case of a 36 year-old female with a primary mediastinal germ cell tumor mimicking an ectopic pregnancy. The patient under birth control pill presented, at seven weeks of amenorrhea, a β-human chorionic gonadotropin (β-hCG) level of 850 UI and uterine vacuity with left lateral uterine heterogeneous mass but no bleeding and no pain. She received left adnexectomy, uterine curettage and further treatment by methotrexate because of persistent high β-hCG markers. Computed tomography scan finally permitted to discover a voluminous anterior mediastinal tumor. We may recommend investigating patients with a simple chest X-ray that present with persistent increased β-hCG despite efficient ectopic pregnancy treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3254848/

Record 1430

pmcid: PMC3739552

pmid: 21892196

doi: 10.1038/aja.2011.86

title: A systematic review of the effects and mechanisms of preoperative 5α-reductase inhibitors on intraoperative haemorrhage during surgery for benign prostatic hyperplasia.

authors

- Zong HT
- Peng XX
- Yang CC
- Zhang Y

journal: Asian journal of andrology

journalAbbr: Asian J Androl

year: 2011

pubdate: 2011 Nov

volume: 13

issue: 6

pages: 812-8

abstract: 5α-reductase inhibitors (5α-RIs), including finasteride and dutasteride, are commonly used medical therapies for benign prostatic hyperplasia (BPH). Many studies reported that preoperative 5α-RI had impact on intraoperative haemorrhage during surgery for BPH, but it was still in controversial. So, we conducted a systematic review of the effects and mechanisms of 5α-RIs on intraoperative bleeding for BPH. MEDLINE, EMBASE, the Cochrane Controlled Trail Register of Controlled Trials and the reference lists of retrieved studies were searched in the analysis. Sixteen publications involving 15 different randomized controlled trials (RCTs) and a total of 1156 patients were used in the analysis, including 10 RCTs for finasteride and five RCTs for dutasteride. We found that preoperative finasteride treatment decreases microvessel density (MVD) in resected prostate specimens. Total blood loss, blood loss per gram of resected prostate tissue and decreases in haemoglobin were all greatly reduced in the finasteride group as compared to controls. Dutasteride appeared to have no effect on bleeding. This meta-analysis shows that preoperative finasteride treatment could decrease intraoperative haemorrhage during surgery for BPH. Preoperative dutasteride had no effect on intraoperative haemorrhage, but further high-quality prospective studies are still needed to confirm this observation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3739552/

Record 1431

pmcid: PMC3312147

pmid: 22547970

doi: 10.1055/s-0031-1281482

title: Aesthetic surgery of the female genitalia.

authors

- Dobbeleir JM
- Landuyt KV
- Monstrey SJ

journal: Seminars in plastic surgery

journalAbbr: Semin Plast Surg

year: 2011

pubdate: 2011 May

volume: 25

issue: 2

pages: 130-41

abstract: Aesthetic genital surgery seems to have become a fashionable issue nowadays. Many procedures and techniques have been described these last years, but very few long-term results or follow up studies are available. The novelty of this aspect of plastic surgery and the lack of evidence-based interventions, have led to a comparison with female genital mutilation. In this article, the authors provide an overview of the possible surgical procedures as well as the general principles of aesthetic surgery of the female genitalia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3312147/

Record 1432

pmcid: PMC3169870

pmid: 21966653

doi: 10.4103/2230-8210.83356

title: An unusual presentation of a usual disorder: Van Wyk-Grumbach syndrome.

authors

- Rastogi A
- Bhadada SK
- Bhansali A

journal: Indian journal of endocrinology and metabolism

journalAbbr: Indian J Endocrinol Metab

year: 2011

pubdate: 2011 Jul

volume: 15

issue: Suppl 2

pages: S141-3

abstract: Van Wyk-Grumbach syndrome (VWGS) is characterized by juvenile hypothyroidism, delayed bone age, and isosexual precocious puberty. A 10.7-year-old girl presented with premature menarche and isosexual precocity. She had delayed bone age and multicystic ovaries. High circulating levels of TSH with prepubertal LH confirmed the diagnosis of VWGS. Tendency to manifest sexual precocity in VWGS may be directly related to the severity of the TSH elevation. High circulating levels of TSH acting directly on FSH receptors are the actual mediators of precocity.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3169870/

Record 1433

pmcid: PMC3265289

pmid: 22312573

doi: 10.4061/2011/703730

title: Artemether-Lumefantrine Combination Therapy for Treatment of Uncomplicated Malaria: The Potential for Complex Interactions with Antiretroviral Drugs in HIV-Infected Individuals.

authors

- Byakika-Kibwika P
- Lamorde M
- Mayanja-Kizza H
- Khoo S
- Merry C
- Van Geertruyden JP

journal: Malaria research and treatment

journalAbbr: Malar Res Treat

year: 2011

pubdate: 2011

volume: 2011

issue:

pages: 703730

abstract: Treatment of malaria in HIV-infected individuals receiving antiretroviral therapy (ART) poses significant challenges. Artemether-lumefantrine (AL) is one of the artemisisnin-based combination therapies recommended for treatment of malaria. The drug combination is highly efficacious against sensitive and multidrug resistant falciparum malaria. Both artemether and lumefantrine are metabolized by hepatic cytochrome P450 (CYP450) enzymes which metabolize the protease inhibitors (PIs) and nonnucleoside reverse transcriptase inhibitors (NNRTIs) used for HIV treatment. Coadministration of NNRTIs and PIs with AL could potentially cause complex pharmacokinetic drug interactions. NNRTI by inducing CYP450 3A4 enzyme and PIs by inhibiting CYP450 3A4 enzymes could influence both artemether and lumefantrine concentrations and their active metabolites dihydroartemisinin and desbutyl-lumefantrine, predisposing patients to poor treatment response, toxicity, and risk for development of resistance. There are scanty data on these interactions and their consequences. Pharmacokinetic studies to evaluate these interactions in the target populations are urgently needed.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3265289/

Record 1434

pmcid: PMC3192272

pmid: 21455827

doi: 10.1007/s00266-011-9680-5

title: Awareness of breast developmental anomalies: a study in Jamasi, Ghana.

authors

- Agbenorku P
- Agbenorku M
- Iddi A
- Amevor E
- Kofitse M
- Klutsey E

journal: Aesthetic plastic surgery

journalAbbr: Aesthetic Plast Surg

year: 2011

pubdate: 2011 Oct

volume: 35

issue: 5

pages: 745-9

abstract: BACKGROUND: Few global studies investigating breast developmental anomalies (BDA) among young females have been conducted. This study aimed to evaluate the degree of BDA awareness among young females in central Ghana. METHODS: In February 2008, clinical breast examination was performed for both breasts of female volunteers at five selected junior high schools (JHS) in Jamasi, Ghana. Anonymous pretested questionnaires were administered to those found to have BDA. RESULTS: Of the 600 female students surveyed, 78 (13%) were found to have BDA. All 78 females completed the questionnaire. The respondents ranged in age from 12 to 21 years (mean 15.3 years). Most of the respondents (83%) had some knowledge of BDA. Although 63% had been aware of their own BDA for 1-3 years before the survey; 78% did not know the cause of their BDA. According to their self-reports, BDA had no influence on the schooling of 78.2% or the family of 70.5% of the respondents. CONCLUSION: Awareness of BDA among the JHS females in Jamasi is high. Appropriate education and treatment should be encouraged for continual management of a high awareness level for BDA.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3192272/

Record 1435

pmcid: PMC3062123

pmid: 21455413

doi: 10.4103/0253-7613.75657

title: Benign prostatic hyperplasia: An overview of existing treatment.

authors

- Dhingra N
- Bhagwat D

journal: Indian journal of pharmacology

journalAbbr: Indian J Pharmacol

year: 2011

pubdate: 2011 Feb

volume: 43

issue: 1

pages: 6-12

abstract: Benign prostatic hyperplasia (BPH) is the most common condition in aging men, associated with lower urinary tract symptoms (LUTS). A better understanding of the prostate physiology, function, and pathogenesis has led to the development of promising agents, useful in the management of LUTS in men. The specific approach used to treat BPH depends upon number of factors like age, prostrate size, weight, prostate-specific antigen level, and severity of the symptoms. 5α-reductase inhibitors decrease the production of dihydrotestosterone within the prostate, which results in decreased prostate volume, increased peak urinary flow rate, improvement of symptoms, decreased risk of acute urinary retention, and need for surgical intervention. α(1)-adrenergic receptor (α(1)-AR) antagonists decrease LUTS and increase urinary flow rates in men with symptomatic BPH, but do not reduce the long-term risk of urinary retention or need for surgical intervention. Clinical efficacy of either 5α-reductase inhibitor or α(1)-AR antagonist has been further improved by using combination therapy; however, long-term outcomes are still awaited. Many more potential new therapies are under development that may improve the treatment of BPH. This article gives a brief account of rationale and efficacy of different treatment options presently available in the management of BPH.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3062123/

Record 1436

pmcid: PMC3207732

pmid: 22675084

doi: 10.1136/bcr.01.2011.3761

title: Bloody nipple discharge in a 7-month-old boy.

authors

- Zaid A
- Carolan E
- Sharif F

journal: BMJ case reports

journalAbbr: BMJ Case Rep

year: 2011

pubdate: 2011 Oct 28

volume: 2011

issue:

pages:

abstract: Isolated bloody nipple discharge is rare in infancy and is usually idiopathic. Discharge commonly resolves spontaneously, and ultrasonography is a useful diagnostic technique to detect the cause of discharge. The authors report a 7-month-old boy who presented with unilateral spontaneous bloody nipple discharge for 1 month without signs of infection or mass.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3207732/

Record 1437

pmcid: PMC3062133

pmid: 21455433

doi: 10.4103/0253-7613.75681

title: Breast enlargement associated with low dose olanzapine.

authors

- Aggarwal A
- Khandelwal A

- Jain M
- Jiloha RC

journal: Indian journal of pharmacology

journalAbbr: Indian J Pharmacol

year: 2011

pubdate: 2011 Feb

volume: 43

issue: 1

pages: 87-8

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3062133/

Record 1438

pmcid: PMC3237339

pmid: 21118785

doi: 10.1289/ehp.1002676

title: Current assessment of the effects of environmental chemicals on the mammary gland in guideline rodent studies by the U.S. Environmental Protection Agency (U.S. EPA), Organisation for Economic Co-operation and Development (OECD), and National Toxicology Program (NTP).

authors

- Makris SL

journal: Environmental health perspectives

journalAbbr: Environ Health Perspect

year: 2011

pubdate: 2011 Aug

volume: 119

issue: 8

pages: 1047-52

abstract: BACKGROUND: Evaluation of the structural and/or functional integrity of the mammary gland (MG) across life stages is integral to the assessment of developmental, reproductive, and carcinogenic risk for environmental chemicals. OBJECTIVES: In this commentary I characterize MG assessment recommended in U.S. Environmental Protection Agency, Organisation for Economic Co-operation and Development, and National Toxicology Program guideline toxicology study protocols and identify any information gaps for the evaluation of MG development, structure, and function. DISCUSSION: Several data gaps, issues, and challenges were identified. Current guidelines that include a lactation phase do not provide specific recommendations to record observations on maternal or offspring lactation or nursing behavior. In guideline studies, the assessment of MG toxicity often relies upon indirect, nonspecific, or surrogate end points, and information that could be useful in the interpretation of these data (e.g., mode of action or toxicokinetics) is often unavailable. Most guideline studies designed to assess general organ toxicity do not expose test animals during sensitive stages of MG development; histopathological evaluation of the developing MG is not routinely conducted; and evaluation of MG tissue for both sexes is inconsistently recommended. CONCLUSIONS: I propose the following general recommendations to enhance MG assessment in guideline toxicology studies: a) inclusion of more specific criteria for the evaluation of MG end points in guideline language, b) inclusion of histopathological evaluation of MG development (using whole-mount techniques) in existing or new guideline protocols that include offspring with perinatal and/or pubertal treatment, c) incorporation of perinatal exposures

into rodent subchronic and carcinogenicity assays, and d) expansion of the histopathological evaluation of male MG tissue.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3237339/

Record 1439

pmcid: PMC3531556

pmid: 21369831

doi: 10.1007/s10897-011-9350-4

title: Development and evaluation of a decision aid for BRCA carriers with breast cancer.

authors

- Culver JO
- MacDonald DJ
- Thornton AA
- Sand SR
- Grant M
- Bowen DJ
- Burke H
- Garcia N
- Metcalfe KA
- Weitzel JN

journal: Journal of genetic counseling

journalAbbr: J Genet Couns

year: 2011

pubdate: 2011 Jun

volume: 20

issue: 3

pages: 294-307

abstract: BRCA+ breast cancer patients face high risk for a second breast cancer and ovarian cancer. Helping these women decide among risk-reducing options requires effectively conveying complex, emotionally-laden, information. To support their decision-making needs, we developed a web-based decision aid (DA) as an adjunct to genetic counseling. Phase 1 used focus groups to determine decision-making needs. These findings and the Ottawa Decision Support Framework guided the DA development. Phase 2 involved nine focus groups of four stakeholder types (BRCA+ breast cancer patients, breast cancer advocates, and genetics and oncology professionals) to evaluate the DA's decision-making utility, information content, visual display, and implementation. Overall, feedback was very favorable about the DA, especially a values and preferences ranking-exercise and an output page displaying personalized responses. Stakeholders were divided as to whether the DA should be offered at-home versus only in a clinical setting. This well-received DA will be further tested to determine accessibility and effectiveness.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3531556/

Record 1440

pmcid: PMC3017875

pmid: 21228947

doi: 10.3348/kjr.2011.12.1.113

title: Diffuse infiltrative lesion of the breast: clinical and radiologic features.

authors

- An YY
- Kim SH
- Cha ES
- Kim HS
- Kang BJ
- Park CS
- Jung NY
- Whang IY
- Yoon SK

journal: Korean journal of radiology

journalAbbr: Korean J Radiol

year: 2011

pubdate: 2011 Jan-Feb

volume: 12

issue: 1

pages: 113-21

abstract: The purpose of this paper is to show the clinical and radiologic features of a variety of diffuse, infiltrative breast lesions, as well to review the relevant literature. Radiologists must be familiar with the various conditions that can diffusely involve the breast, including normal physiologic changes, benign disease and malignant neoplasm.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3017875/

Record 1441

pmcid: PMC3182960

pmid: 21929773

doi: 10.1186/1746-1596-6-84

title: Dysgerminoma in a case of 46, XY pure gonadal dysgenesis (Swyer syndrome): a case report.

authors

- Han Y
- Wang Y
- Li Q
- Dai S
- He A
- Wang E

journal: Diagnostic pathology

journalAbbr: Diagn Pathol

year: 2011

pubdate: 2011 Sep 19

volume: 6

issue:

pages: 84

abstract: Simple 46, XY gonadal dysgenesis syndrome, also called Swyer syndrome, is known as pure gonadal dysgenesis. Individuals with the syndrome are characterized by 46, XY karyotype and phenotypically female with female genital appearance, normal Müllerian structures and absent testicular tissue. The condition usually first becomes apparent in adolescence with delayed puberty and primary amenorrhea due to the gonads have no hormonal or reproductive potential. Herein, we report a case of dysgerminoma diagnosed in a dysgenetic gonad of a 21-year-old patient with Swyer syndrome.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3182960/

Record 1442

pmcid: PMC3059593

pmid: 21239023

doi: 10.1016/j.juro.2010.10.078

title: Effects of family history and genetic polymorphism on the cost-effectiveness of chemoprevention with finasteride for prostate cancer.

authors

- Reed SD
- Scales CD Jr
- Stewart SB
- Sun J
- Moul JW
- Schulman KA
- Xu J

journal: The Journal of urology

journalAbbr: J Urol

year: 2011

pubdate: 2011 Mar

volume: 185

issue: 3

pages: 841-7

abstract: PURPOSE: Improvement in the cost-effectiveness of chemoprevention for prostate cancer could be realized through the identification of patients at higher risk. We estimated the cost-effectiveness of prostate cancer chemoprevention across risk groups defined by family history and number of risk alleles, and the cost-effectiveness of targeting chemoprevention to higher risk groups. MATERIALS AND METHODS: We developed a probabilistic Markov model to estimate costs, survival and quality adjusted survival across risk groups for patients receiving or not receiving chemoprevention with finasteride. The model uses data from national cancer registries, online sources and the medical literature. RESULTS: The incremental cost-effectiveness of 25 years of chemoprevention with finasteride in patients 50 years old was an estimated $89,300 per quality adjusted life-year (95% CI $58,800-$149,800), assuming finasteride decreased all grades of prostate cancer by 24.8%. Among patients with a positive family history (without genetic testing) chemoprevention provided 1 additional quality adjusted life-year at a cost of $64,200. Among patients with a negative family history at $400 per person tested, the cost-effectiveness of genetically targeted chemoprevention ranged from $98,100 per quality adjusted life-year when limiting finasteride to individuals with 14 or more risk alleles, to $103,200 per quality adjusted life-year when including those with 8 or more risk alleles. CONCLUSIONS: Although there are small differences in the cost-effectiveness of genetically targeted chemoprevention strategies in patients with a negative family

history, genetic testing could reduce total expenditures if used to target chemoprevention for higher risk groups.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3059593/

Record 1443

pmcid: PMC5044559

pmid: 22506441

doi: 10.64719/pb.4073

title: Euprolactinemic Gynecomastia and Galactorrhea with Risperidone-Fluvoxamine Combination.

authors

- P J P
- Praharaj SK
- Srivastava A

journal: Psychopharmacology bulletin

journalAbbr: Psychopharmacol Bull

year: 2011

pubdate: 2011 Feb 15

volume: 44

issue: 1

pages: 70-73

abstract: Risperidone is associated with hyperprolactinemia and its consequent symptoms such as gynnecomastia, galactorrhea and sexual dysfunction in adults, and less so in adolescents. Rarely, serotonin reuptake inhibitors are also associated with such adverse effects. We report a case of gynecomastia and galactorrhea in an adolescent male while on a combination of risperidone and fluvoxamine, although the serum prolactin was within normal range.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5044559/

Record 1444

pmcid: PMC7271204

pmid:

doi: 10.1016/B978-1-4160-6400-8.00010-9

title: Focal Bacterial Infections.

authors

- Overturf GD

journal: Infectious Diseases of the Fetus and Newborn

journalAbbr: Infectious Diseases of the Fetus and Newborn

year: 2011

pubdate: 2011

volume:

issue:

pages: 322-58

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7271204/

Record 1445

pmcid: PMC3062818

pmid: 22707463

doi: 10.1136/bcr.01.2010.2632

title: Gestational gigantomastia with complete resolution in a Nigerian woman.

authors

- Ezem BU
- Osuagwu CC
- Opara KA

journal: BMJ case reports

journalAbbr: BMJ Case Rep

year: 2011

pubdate: 2011 Feb 15

volume: 2011

issue:

pages:

abstract: Physiological enlargement of the breasts occurs at puberty and during pregnancy. It is known as gestational gigantomastia when enlargement in pregnancy becomes excessive, uncomfortable and embarrassing. Gestational gigantomastia may have far reaching effects for the mother and fetus. This rare condition is associated with considerable morbidity but may be associated with good fetal outcome. Our case was very interesting because it was managed conservatively and postpartum there was complete spontaneous resolution.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3062818/

Record 1446

pmcid: PMC3174867

pmid: 21864353

doi: 10.1186/1472-6874-11-39

title: Group medical visits in the follow-up of women with a BRCA mutation: design of a randomized controlled trial.

authors

- Visser A
- Prins JB
- Hoogerbrugge N
- van Laarhoven HW

journal: BMC women's health

journalAbbr: BMC Womens Health

year: 2011

pubdate: 2011 Aug 24

volume: 11

issue:

pages: 39

abstract: BACKGROUND: BRCA mutation carriers have a 40-80% life-time risk of developing breast cancer. They may opt for yearly breast cancer surveillance or for prophylactic mastectomy. Both options show increased survival rates. It is a complex choice to be made between these two options. As a result most women experience high levels of distress and high needs for information. To fulfill the needs for psychosocial support and information we have introduced group medical consultations (GMCs). A GMC provides individual medical visits conducted within a group. This 90 minute group-visit with 8-12 patients gives patients the opportunity to spend more time with their clinician and a behavioral health professional and learn from other patients experiencing similar topics. However, it should be noted that group sessions may increase fear in some patients or influence their decision making. METHODS/DESIGN: In this randomized controlled trial, 160 BRCA mutation carriers diagnosed maximally 2 years ago are recruited from the Radboud University Nijmegen Medical Centre. Participants are randomized in a 1:1 ratio to either the GMC intervention group (onetime participation in a GMC instead of a standard individual visit) or to a usual care control group. Primary outcome measures are empowerment and psychological distress (SCL 90). Secondary outcome measures are fear of cancer, information needs before the consultation and the received information, self-examination of the breasts, patient satisfaction, quality of life and cost-effectiveness. Data are collected via self-reported questionnaires 1 week before the visit, and at 1 week and at 3 months follow-up. A pilot study was conducted to test all procedures and questionnaires. DISCUSSION: The possibility for interaction with other BRCA mutation carriers within a medical visit is unique. This study will assess the effectiveness of GMCs for BRCA mutation carriers to improve empowerment and decrease distress compared to individual visits. If GMCs prove to be effective and efficient, implementation of GMCs in regular care for BRCA mutation carriers will be recommended.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3174867/

Record 1447

pmcid: PMC3146708

pmid: 21136254

doi: 10.1007/s00266-010-9629-0

title: How important is "pseudogynecomastia"?

authors

- Venkata Ratnam B

journal: Aesthetic plastic surgery

journalAbbr: Aesthetic Plast Surg

year: 2011

pubdate: 2011 Aug

volume: 35

issue: 4

pages: 668-9

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3146708/

Record 1448

pmcid: PMC3193773

pmid: 22028995

doi: 10.4103/2230-8210.85574

title: Human immunodeficiency virus endocrinopathy.

authors

- Sinha U
- Sengupta N
- Mukhopadhyay P
- Roy KS

journal: Indian journal of endocrinology and metabolism

journalAbbr: Indian J Endocrinol Metab

year: 2011

pubdate: 2011 Oct

volume: 15

issue: 4

pages: 251-60

abstract: Human immunodeficiency virus (HIV) endocrinopathy encompasses a broad spectrum of disorders. Almost all the endocrine organs are virtually affected by HIV infection. HIV can directly alter glandular function. More commonly secondary endocrine dysfunction occurs due to opportunistic infections and neoplasms in immunocompromised state. The complex interaction between HIV infection and endocrine system may be manifested as subtle biochemical and hormonal perturbation to overt glandular failure. Antiretroviral therapy as well as other essential medications often result in adverse endocrinal consequences. Apart from adrenal insufficiency, hypogonadism, diabetes and bone loss, AIDS wasting syndrome and HIV lipodystrophy need special reference. Endocrinal evaluation should proceed as in other patients with suspected endocrine dysfunction. Available treatment options have been shown to improve quality of life and long-term mortality in AIDS patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3193773/

Record 1449

pmcid: PMC3229802

pmid: 22144791

doi: 10.4103/0253-7613.89843

title: Hyperprolactinemia and breast enlargement with serotonin reuptake inhibitor and olanzapine combination.

authors

- Praharaj SK

journal: Indian journal of pharmacology

journalAbbr: Indian J Pharmacol

year: 2011

pubdate: 2011 Nov

volume: 43

issue: 6

pages: 739-40

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3229802/

Record 1450

pmcid: PMC6264586

pmid: 22138857

doi: 10.3390/molecules16129983

title: Inhibitors of testosterone biosynthetic and metabolic activation enzymes.

authors

- Ye L
- Su ZJ
- Ge RS

journal: Molecules (Basel, Switzerland)

journalAbbr: Molecules

year: 2011

pubdate: 2011 Dec 2

volume: 16

issue: 12

pages: 9983-10001

abstract: The Leydig cells of the testis have the capacity to biosynthesize testosterone from cholesterol. Testosterone and its metabolically activated product dihydrotestosterone are critical for the development of male reproductive system and spermatogenesis. At least four steroidogenic enzymes are involved in testosterone biosynthesis: Cholesterol side chain cleavage enzyme (CYP11A1) for the conversion of cholesterol into pregnenolone within the mitochondria, 3β-hydroxysteroid dehydrogenase (HSD3B), for the conversion of pregnenolone into progesterone, 17α-hydroxylase/17,20-lyase (CYP17A1) for the conversion of progesterone into androstenedione and 17β-hydroxysteroid dehydrogenase (HSD17B3) for the formation of testosterone from androstenedione. Testosterone is also metabolically activated into more potent androgen dihydrotestosterone by two isoforms 5α-reductase 1 (SRD5A1) and 2 (SRD5A2) in Leydig cells and peripheral tissues. Many endocrine disruptors act as antiandrogens via directly inhibiting one or more enzymes for testosterone biosynthesis and metabolic activation. These chemicals include industrial materials (perfluoroalkyl compounds, phthalates, bisphenol A and benzophenone) and pesticides/biocides (methoxychlor, organotins, 1,2-dibromo-3-chloropropane and prochloraz) and plant constituents (genistein and gossypol). This paper reviews these endocrine disruptors targeting steroidogenic enzymes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6264586/

Record 1451

pmcid: PMC3269333

pmid: 22942662

doi: 10.1177/229255031101900308

title: Juvenile fibroadenoma of the breast: Treatment and literature review.

authors

- Ng WK
- Mrad MA
- Brown MH

journal: The Canadian journal of plastic surgery = Journal canadien de chirurgie plastique

journalAbbr: Can J Plast Surg

year: 2011

pubdate: 2011 Fall

volume: 19

issue: 3

pages: 105-7

abstract: It is uncommon to find palpable breast masses in young patients. Generally, such masses are benign. Juvenile fibroadenoma is the most frequent benign tumour of the breast. The present report describes a case involving a 17-year-old girl with unilateral right breast hypertrophy, for whom resection and breast reduction was effectively achieved.Resection was combined with an aesthetically pleasing outcome through breast reduction, via a wise pattern of reduction with a superior pedicle.While young patients rarely present with breast masses, the consideration of fibroadenoma in these cases is always warranted. The disconcerting appearance of the breasts for adolescents afflicted by such benign masses underscores the importance of achieving aesthetically optimal outcomes and attaining adequate tumour resection for diagnostic purposes through pathological specimens. In the present case, the young patient underwent a procedure that fulfilled both of these valuable goals through careful pre-operative planning, pedicle selection and skin resection patterns.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3269333/

Record 1452

pmcid: PMC3251947

pmid: 22048030

doi: 10.1038/bjc.2011.420

title: Long-term health outcomes in a British cohort of breast, colorectal and prostate cancer survivors: a database study.

authors

- Khan NF
- Mant D
- Carpenter L
- Forman D
- Rose PW

journal: British journal of cancer

journalAbbr: Br J Cancer

year: 2011

pubdate: 2011 Nov 8

volume: 105 Suppl 1

issue: Suppl 1

pages: S29-37

abstract: BACKGROUND: The community-based incidence of cancer treatment-related long-term consequences is uncertain. We sought to establish the burden of health outcomes that have been associated with treatment among British long-term cancer survivors. METHODS: We identified 26,213 adults from the General Practice Research Database who have survived 5 years or more following breast, colorectal or prostate cancer. Four age-, sex- and general practice-matched non-cancer controls were selected for each survivor. We considered the incidence of treatment-associated health outcomes using Cox proportional hazards models. RESULTS: Breast cancer survivors had an elevated incidence of heart failure (hazards ratio (HR) 1.95, 95% confidence interval (CI) 1.27-3.01), coronary artery disease (HR 1.27, 95% CI 1.11-1.44), hypothyroidism (HR 1.26, 95% CI 1.02-1.56) and osteoporosis (HR 1.26, 95% CI 1.13-1.40). Among colorectal cancer survivors, there was increased incidence of dementia (HR 1.68, 95% CI 1.20-2.35), diabetes (HR 1.39, 95% CI 1.12-1.72) and osteoporosis (HR 1.41, 95% CI 1.15-1.73). Prostate cancer survivors had the highest risk of osteoporosis (HR 2.49, 95% CI 1.93-3.22). CONCLUSIONS: The study confirms the occurrence of increased incidence of chronic illnesses in long-term cancer survivors attributable to underlying lifestyle and/or cancer treatments. Although the absolute risk of the majority of late effects in the cancer survivors cohort is low, identifying prior risk

of osteoporosis by bone mineral density scanning for prostate survivors should be considered. There is an urgent need to improve primary care recording of cancer treatment.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3251947/

Record 1453

pmcid: PMC3171824

pmid: 21935296

doi:

title: Lurasidone HCl (Latuda), an Oral, Once-Daily Atypical Antipsychotic Agent for the Treatment of Patients with Schizophrenia.

authors

- Cruz MP

journal: P & T : a peer-reviewed journal for formulary management

journalAbbr: P T

year: 2011

pubdate: 2011 Aug

volume: 36

issue: 8

pages: 489-92

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3171824/

Record 1454

pmcid: PMC3265148

pmid: 22287984

doi: 10.2349/biij.7.2.e10

title: Lymphoma presenting as gynaecomastia.

authors

- Mahmood S
- Sabih Z
- Sabih D

journal: Biomedical imaging and intervention journal

journalAbbr: Biomed Imaging Interv J

year: 2011

pubdate: 2011 Apr

volume: 7

issue: 2

pages: e10

abstract: Breast lymphoma is an uncommon neoplasm affecting the breast and is extremely rare in males. While gynaecomastia is common and in most cases benign, it can sometimes result from significant pathology and the physician should keep in mind the possible diseases that can lead to gynaecomastia. This paper reports a case of lymphoma presenting as unilateral gynaecomastia. The paper discusses the differential diagnosis and

emphasises the points that should raise the suspicion of pathology.Mammography, high resolution ultrasound and biopsy findings are discussed and literature survey is presented.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3265148/

Record 1455

pmcid: PMC3171812

pmid: 21931472

doi:

title: Pharmaceutical approval update.

authors

- Goldenberg MM

journal: P & T : a peer-reviewed journal for formulary management

journalAbbr: P T

year: 2011

pubdate: 2011 Jul

volume: 36

issue: 7

pages: 410-51

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3171812/

Record 1456

pmcid: PMC3262531

pmid: 22363845

doi: 10.5402/2011/241953

title: Pilot Study of 15 Patients Receiving a New Treatment Regimen for Androgenic Alopecia: The Effects of Atopy on AGA.

authors

- Rafi AW
- Katz RM

journal: ISRN dermatology

journalAbbr: ISRN Dermatol

year: 2011

pubdate: 2011

volume: 2011

issue:

pages: 241953

abstract: Background. We examined the efficacy of a new regimen to treat AGA, with attention to male patients who are atopic. Objective. To assess the efficacy of a four-part regimen for the treatment of AGA in atopic and nonatopic patients. NuH Hair is a novel topical combination of finasteride, dutasteride, and minoxidil, which is blended in a hypoallergenic lotion. The other three components included Rogaine foam, Propecia, and ketoconazole shampoo. Methods. A prospective pilot study was conducted in 15 patients. All patients were

assessed for the presence of atopy. Each patient served as their own control. All patients were treated specifically with NuH Hair and were given the option to add any of the other components of the protocol to their regimen. Photographs were taken of each patient's scalp at months 0, 1, 3, 6, and 9. Results. All 15 patients demonstrated significant growth of hair. In those patients who utilized all 4 components, significant growth was achieved in as little as 30 days. In those patients who choose only to utilize NuH Hair, significant growth was demonstrated after 3 months. Conclusion. Aggressively treating AGA achieves significant and rapid growth of new hair. This is effective in atopic and nonatopic male patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3262531/

Record 1457

pmcid: PMC3420754

pmid: 22937289

doi: 10.1155/2011/678305

title: Premature menarche associated with primary hypothyroidism in a 5.5-year-old girl.

authors

- Sharma D
- Dayal D
- Gupta A
- Saxena A

journal: Case reports in endocrinology

journalAbbr: Case Rep Endocrinol

year: 2011

pubdate: 2011

volume: 2011

issue:

pages: 678305

abstract: Children with hypothyroidism generally have delayed pubertal development. Rare association with precocious puberty may occur especially in long standing untreated patients. The cardinal features of hypothyroidism induced pseudo precocious pubertal development include thelarche, galactorrhea and menarche. Other characteristics features are an absence of sexual hair and retardation of linear growth. Its manifestation as isolated menarche has been rarely reported. Recently, a five and half year old girl presented to us with history of one episode of vaginal bleeding. A pelvic ultrasonogram revealed multiple cysts in both ovaries and subsequent investigations led to a diagnosis of autoimmune hypothyroidism.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3420754/

Record 1458

pmcid: PMC3178024

pmid: 21966096

doi: 10.1007/s00238-010-0469-6

title: Prepubertal unilateral gynecomastia: a report of two cases.

authors

- Hoevenaren IA
- Schott DA
- Otten BJ

- Kroese-Deutman HC

journal: European journal of plastic surgery

journalAbbr: Eur J Plast Surg

year: 2011

pubdate: 2011 Oct

volume: 34

issue: 5

pages: 395-398

abstract: BACKGROUND: Gynecomastia is defined as the presence of excessive breast tissue in males, which can appear unilateral or bilateral. Bilateral gynecomastia is frequently found in the neonatal period, early in puberty, and with increasing age. Prepubertal unilateral gynecomastia in the absence of endocrine abnormalities is extremely rare, with only a few cases in literature. METHODS: We report the cases of two otherwise healthy boys of 8 and 11 years old with unilateral breast masses. No abnormalities were found on ultrasonography and all endocrine parameters were within normal limits. Treatment consisted of peripheral liposuction followed by subcutaneous partial resection of the gland, conducted through an infra-areolar incision. RESULTS: Microscopy of the subcutaneous mastectomy specimen revealed gynecomastia without signs of malignancy. Postoperative course of both patients was uncomplicated, with no signs of recurrence of breast tissue. CONCLUSIONS: Atypical presentations of gynecomastia are often not recognized, with little attention to breast development in prepubertal non-obese children. Since prepubertal gynecomastia could be a sign of possible underlying diseases, a thorough examination and further research is recommended. If there is no causal treatment, surgical resection is the therapy of first choice. Peripheral liposuction and surgical resection of the gland tissue are the mainstay of treatment. In summary, we describe two cases of prepubertal unilateral gynecomastia with a normal endocrine workup. Further research is needed to establish the pathophysiologic mechanisms of prepubertal gynecomastia, since underlying etiology in most cases remains unclear.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3178024/

Record 1459

pmcid: PMC3522409

pmid: 23329915

doi:

title: Primary non-hodgkin B cell lymphoma in a man.

authors

- Alhabshi ShM
- Ismail Z
- Arasaratnam ShA

journal: Iranian journal of radiology : a quarterly journal published by the Iranian Radiological Society

journalAbbr: Iran J Radiol

year: 2011

pubdate: 2011 Mar

volume: 8

issue: 1

pages: 39-41

abstract: Malignant breast lymphoma is a rare condition and primary breast lymphoma is extremely rare in the male population. We present a case of a 26-year-old man (transgender) who presented with a large palpable mass in the right breast. This mass was rapidly growing in size associated with right axillary lymphadenopathy.

Ultrasound and MRI findings were consistent with BIRADS IV lesion which was suspicious of malignancy. Core biopsy was performed and histopathology confirmed the diagnosis of primary non Hodgkin B cell lymphoma of the breast.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3522409/

Record 1460

pmcid: PMC3448126

pmid: 23289044

doi: 10.4066/AMJ.2011.534

title: Psycho-Social Behaviour of Urban Indian Adolescent Girls during menstruation.

authors

- Goel MK
- Kundan M

journal: The Australasian medical journal

journalAbbr: Australas Med J

year: 2011

pubdate: 2011

volume: 4

issue: 1

pages: 49-52

abstract: BACKGROUND: Adolescence in females signifies the transition from girlhood to womanhood and is marked with the onset of menarche. Indian society is interwoven into a set of traditions, myths and misconceptions, especially regarding menstruation and related issues. The present study was conducted to assess knowledge and psycho-social behavior related to menstruation among adolescent girls in urban Haryana (state), India. METHOD: A total of 478 adolescent girls in the age group of 15 -19 years from three educational institutes of Rohtak city were selected randomly. It was a community-based, descriptive, cross-sectional questionnaire based study, and a pre-tested, pre-coded, closed ended questionnaire was used. RESULTS: Feeling of sickness was the most common (in more than two-third of subjects) followed by irritability and emotional disturbances. More than 3/4(th) of the subjects did not worship during menstruation, 45% were not allowed in kitchen and nearly one-fourth followed dietary restrictions. More than 16% subjects thought menstruation to be a sign of onset of a disease and little more than 7 % thought it to be a curse. Girls preferred to discuss their menstruation related problems either with their mothers or with their friends. CONCLUSION: Girls have inaccurate and partial information regarding menstruation. There is a need of early intervention in the area of adolescent psycho-social behavior during menstruation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3448126/

Record 1461

pmcid: PMC3065004

pmid: 21461391

doi: 10.4061/2011/143471

title: Role of the kidneys in resistant hypertension.

authors

- Khawaja Z
- Wilcox CS

journal: International journal of hypertension

journalAbbr: Int J Hypertens

year: 2011

pubdate: 2011 Mar 14

volume: 2011

issue:

pages: 143471

abstract: Resistant hypertension is a failure to achieve goal BP (<140/90 mm Hg for the overall population and <130/80 mm Hg for those with diabetes mellitus or chronic kidney disease) in a patient who adheres to maximum tolerated doses of 3 antihypertensive drugs including a diuretic. The kidneys play a critical role in long-term regulation of blood pressure. Blunted pressure natriuresis, with resultant increase in extracellular fluid volume, is an important cause of resistant hypertension. Activation of the renin-angiotensin-aldosterone system, increased renal sympathetic nervous system activity and increased sodium reabsorption are important renal mechanisms. Successful treatment requires identification and reversal of lifestyle factors or drugs contributing to treatment resistance, diagnosis and appropriate treatment of secondary causes of hypertension, use of effective multidrug regimens and optimization of diuretic therapy. Since inappropriate renal salt retention underlies most cases of drug-resistant hypertension, the therapeutic focus should be on improving salt depleting therapy by assessing and, if necessary, reducing dietary salt intake, optimizing diuretic therapy, and adding a mineralocorticoid antagonist if there are no contraindications.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3065004/

Record 1462

pmcid: PMC3663611

pmid: 23861636

doi: 10.4137/JCNSD.S1607

title: Safety and efficacy of paliperidone extended-release in acute and maintenance treatment of schizophrenia.

authors

- Spina E
- Crupi R

journal: Journal of central nervous system disease

journalAbbr: J Cent Nerv Syst Dis

year: 2011

pubdate: 2011

volume: 3

issue:

pages: 27-41

abstract: Paliperidone, the major active metabolite of risperidone, is a second-generation antipsychotic that has been developed as an extended-release (ER) tablet formulation that minimizes peak-trough fluctuations in plasma concentrations, allowing once-daily administration and constant drug delivery. Paliperidone ER has demonstrated efficacy in the reduction of acute schizophrenia symptoms in 6-week, placebo-controlled, double-blind trials and clinical benefits were maintained in the longer-term according to extension studies of up to 52 weeks in duration. Compared with quetiapine, paliperidone ER was associated with a more rapid symptom improvement. In addition, it was more effective than placebo in the prevention of symptom recurrence. Paliperidone ER is generally well tolerated with a predictable adverse event profile. Like risperidone, it is associated with a dose-dependent risk of extrapyramidal symptoms and prolactin elevation. Short- and longer-term studies have indicated a low liability for paliperidone ER to cause metabolic (ie, weight gain, hyperglycaemia and lipid dysregulation) or cardiovascular adverse effects. Available safety data from elderly

patients appear to be promising. Due to negligible hepatic biotransformation, paliperidone ER is unlikely to be involved in clinically significant metabolic drug-drug interactions. Additional active comparator trials evaluating efficacy, tolerability and cost-effectiveness are required to better define the role of paliperidone ER in the treatment of schizophrenia in relation to other currently available second-generation antipsychotics, particularly risperidone.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3663611/

Record 1463

pmcid: PMC3207633

pmid: 22053162

doi: 10.3402/qhw.v6i4.7378

title: Torrenting values, feelings, and thoughts-Cyber nursing and virtual self-care in a breast augmentation forum.

authors

- Martin Salzmann-Erikson RN
- Henrik Eriksson RN

journal: International journal of qualitative studies on health and well-being

journalAbbr: Int J Qual Stud Health Well-being

year: 2011

pubdate: 2011

volume: 6

issue: 4

pages:

abstract: Earlier research shows that breast augmentation is positively correlated with positive psychological states. The aim of this study was to explore the shared values, feelings, and thoughts within the culture of breast enlargement among women visiting Internet-based forums when considering and/or undergoing esthetic plastic surgery. The study used a netnographic method for gathering and analyzing data. The findings show that the women used the Internet forum to provide emotional support to other women. Through electronic postings, they cared for and nursed each others' anxiety and feelings throughout the whole process. Apart from the process, another central issue was that the women's relationships were frequently discussed; specifically their relationship to themselves, their environment, and with the surgeons. The findings suggest that Internet forums represent a channel through which posters can share values, feelings, and thoughts from the position of an agent of action as well as from a position as the object of action. These dual positions and the medium endow the women with a virtual nursing competence that would otherwise be unavailable. By introducing the concept of torrenting as a means of sharing important self-care information, the authors provide a concept that can be further explored in relation to post modern self-care strategies within contemporary nursing theories and practice.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3207633/

Record 1464

pmcid: PMC3558246

pmid: 23397020

doi: 10.1016/j.jus.2011.06.004

title: Ultrasonography of the male breast.

authors

- Draghi F

- Tarantino CC
- Madonia L
- Ferrozzi G

journal: Journal of ultrasound

journalAbbr: J Ultrasound

year: 2011

pubdate: 2011 Sep

volume: 14

issue: 3

pages: 122-9

abstract: The male breast has been insufficiently explored in the medical literature, particularly that dealing with ultrasonography, although this topic is almost as vast and varied as that of the female breast. The purpose of this article is to provide a schematic review of the most frequent breast lesions encountered in males and their sonographic appearances. After a brief introduction on the anatomy of the male breast, the authors review the non-neoplastic (gynecomastia, pseudogynecomastia, cysts, inflammatory diseases, and Mondor disease) and neoplastic (benign and malignant) lesions encountered in this organ.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3558246/

Record 1465

pmcid: PMC3312601

pmid: 21975003

doi: 10.1016/j.scitotenv.2011.09.029

title: Urinary mycoestrogens, body size and breast development in New Jersey girls.

authors

- Bandera EV
- Chandran U
- Buckley B
- Lin Y
- Isukapalli S
- Marshall I
- King M
- Zarbl H

journal: The Science of the total environment

journalAbbr: Sci Total Environ

year: 2011

pubdate: 2011 Nov 15

volume: 409

issue: 24

pages: 5221-7

abstract: BACKGROUND: Despite extensive research and interest in endocrine disruptors, there are essentially no epidemiologic studies of estrogenic mycotoxins, such as zeranol and zearalenone (ZEA). ZEA mycoestrogens are present in grains and other plant foods through fungal contamination, and in animal products (e.g., meat, eggs, dairy products) through deliberate introduction of zeranol into livestock to enhance meat

production, or by indirect contamination of animals through consumption of contaminated feedstuff. Zeranol is banned for use in animal husbandry in the European Union and other countries, but is still widely used in the US. Surprisingly, little is known about the health effects of these mycoestrogens, including their impact on puberty in girls, a period highly sensitive to estrogenic stimulation. OBJECTIVES AND METHODS: We conducted a cross-sectional analysis among 163 girls, aged 9 and 10 years, participating in the Jersey Girl Study to measure urinary mycoestrogens and their possible relationship to body size and development. RESULTS: We found that mycoestrogens were detectable in urine in 78.5% of the girls, and that urinary levels were predominantly associated with beef and popcorn intake. Furthermore, girls with detectable urinary ZEA mycoestrogen levels tended to be shorter and less likely to have reached the onset of breast development. CONCLUSIONS: Our findings suggest that ZEA mycoestrogens may exert anti-estrogenic effects similar to those reported for isoflavones. To our knowledge, this was the first evaluation of urinary mycoestrogens and their potential health effects in healthy girls. However, our findings need replication in larger studies with more heterogeneous populations, using a longitudinal approach.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3312601/

Record 1466

pmcid: PMC3005665

pmid: 21274335

doi: 10.4274/jcrpe.v2i1.39

title: Acquired generalized lipodystrophy associated with autoimmune hepatitis and low serum C4 level.

authors

- Eren E
- Özkan TB
- Çakır ED
- Sağlam H
- Tarım Ö

journal: Journal of clinical research in pediatric endocrinology

journalAbbr: J Clin Res Pediatr Endocrinol

year: 2010

pubdate: 2010

volume: 2

issue: 1

pages: 39-42

abstract: Lipodystrophies are a group of diseases characterized by loss of fat tissue and are associated with insulin resistance. A six-year-old girl followed with the diagnosis of autoimmune hepatitis showed a severe loss of fat tissue, hyperinsulinemia, impaired glucose tolerance, hypertriglyceridemia and low serum complement 4 (C4) levels. She had coarse facial features with generalized loss of subcutaneous fat and prominent muscularity. Remarkable acanthosis nigricans was present over the neck, axilla, and umbilicus. Two hours after glucose loading, the glucose tolerance test revealed a glucose level of 258 mg/dL, a HbA1c value of 6.8%, and an insulin level of 642.9 mIU/mL, documenting a state of insulin resistance and type 2 diabetes mellitus. Acquired generalized lipodystrophy was diagnosed and metformin with dietary intervention was initiated. Low serum complement levels proved the autoimmune nature of the process. We conclude that the serum complement levels must be investigated in patients with acquired lipodystrophy, particularly when it is associated with autoimmune hepatitis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3005665/

Record 1467

pmcid: PMC2957859

pmid: 20981348

doi: 10.1155/2010/708481

title: Anaesthesia for emergency caesarean section in a patient with large anterior mediastinal tumour presenting as intrathoracic airway compression and superior vena cava obstruction.

authors

- Chiang JC
- Irwin MG
- Hussain A
- Tang YK
- Hiong YT

journal: Case reports in medicine

journalAbbr: Case Rep Med

year: 2010

pubdate: 2010

volume: 2010

issue:

pages: 708481

abstract: Anterior mediastinal mass is an uncommon pathology that presents significant anaesthetic challenges because of cardiopulmonary compromise. We present a case that presented in the third trimester of pregnancy with severe breathlessness, orthopnoea, and symptoms of superior vena cava obstruction. The patient had emergency Caesarean section under epidural anaesthesia with a good outcome. The paper discusses the relevant perioperative considerations for this complex scenario and reviews reports of similar conditions.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2957859/

Record 1468

pmcid: PMC2991650

pmid: 21189878

doi: 10.4103/0019-5049.71037

title: Anaesthesia for lower-segment caesarean section: Changing perspectives.

authors

- Yeoh SB
- Leong SB
- Heng AS

journal: Indian journal of anaesthesia

journalAbbr: Indian J Anaesth

year: 2010

pubdate: 2010 Sep

volume: 54

issue: 5

pages: 409-14

abstract: The number of caesarean sections has increased over the last two decades, especially in the developed countries. Hence, it has increasingly become a greater challenge to provide care for the parturient, but this has given obstetric anaesthetists a greater opportunity to contribute to obstetric services. While caesarean deliveries were historically performed using general anaesthesia, there is a recent significant move towards regional anaesthesia. Unique problems that patients with obesity and pre-eclampsia present will be discussed in the present article. New medications and devices now used in obstetric anaesthesia will change the practice and perspectives of our clinical practice.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2991650/

Record 1469

pmcid: PMC3108697

pmid: 21701626

doi: 10.2147/DHPS.S7634

title: Antidepressant-associated sexual dysfunction: impact, effects, and treatment.

authors

- Higgins A
- Nash M
- Lynch AM

journal: Drug, healthcare and patient safety

journalAbbr: Drug Healthc Patient Saf

year: 2010

pubdate: 2010

volume: 2

issue:

pages: 141-50

abstract: Sexual dysfunction is a common side effect of antidepressants and can have significant impact on the person's quality of life, relationships, mental health, and recovery. The reported incidence of sexual dysfunction associated with antidepressant medication varies considerably between studies, making it difficult to estimate the exact incidence or prevalence. The sexual problems reported range from decreased sexual desire, decreased sexual excitement, diminished or delayed orgasm, to erection or delayed ejaculation problems. There are a number of case reports of sexual side effects, such as priapism, painful ejaculation, penile anesthesia, loss of sensation in the vagina and nipples, persistent genital arousal and nonpuerperal lactation in women. The focus of this article is to explore the incidence, pathophysiology, and treatment of antidepressant iatrogenic sexual dysfunction.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3108697/

Record 1470

pmcid: PMC3004507

pmid: 21189964

doi: 10.3345/kjp.2010.53.10.917

title: Bloody nipple discharge in an infant.

authors

- Seo JY
- Kim SJ
- Lee SJ

- Song ES
- Woo YJ
- Choi YY

journal: Korean journal of pediatrics

journalAbbr: Korean J Pediatr

year: 2010

pubdate: 2010 Oct

volume: 53

issue: 10

pages: 917-20

abstract: Although milky nipple discharge appears frequently in infants, bloody nipple discharge is a very rare finding. We experienced a 4-month-old, breast-fed infant who showed bilateral bloody nipple discharge with no signs of infection, engorgement, or hypertrophy. The infant's hormonal examination and coagulation tests were normal, and an ultrasound examination revealed mammary duct ectasia. The symptoms resolved spontaneously within 6 weeks without any specific treatment, except that we advised the mother to refrain from taking herbal medicine. Since no such case has been previously reported in Korea, we present this case with a brief review of the literature.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3004507/

Record 1471

pmcid: PMC2928210

pmid: 20701744

doi: 10.1186/1756-3305-3-70

title: Challenges in mass drug administration for treating lymphatic filariasis in Papua, Indonesia.

authors

- Bhullar N
- Maikere J

journal: Parasites & vectors

journalAbbr: Parasit Vectors

year: 2010

pubdate: 2010 Aug 11

volume: 3

issue:

pages: 70

abstract: BACKGROUND: The World Health Organization (WHO) Global Program to Eliminate Lymphatic Filariasis relies on mass drug administration (MDA) of two drugs annually for 4 to 6 years. The goal is to reduce the reservoir of microfilariae in the blood to a level insufficient to maintain transmission by the mosquito vector. In 2008, the international medical aid organization Médecins Sans Frontières (MSF) performed the first round of a MDA in the high-burden area of Asmat district, in Papua, Indonesia. We report the challenges faced in this MDA on a remote Indonesian island and propose solutions to overcome these hurdles in similar future contexts. RESULTS: During the MDA, we encountered difficult challenges in accessing as well as persuading the patient population to take the antifilarial drugs. Health promotion activities supporting treatment need to be adapted and repetitive, with adequate time and resources allocated for accessing and communicating with local, seminomadic populations. Distribution of bednets resulted in an increase in MDA coverage, but it was still below the 80-85% target. CONCLUSIONS: MDA for lymphatic filariasis is how the WHO has planned to eliminate the

disease from endemic areas. Our programmatic experience will hopefully help inform future campaign planning in difficult-to-access, high-burden areas of the world to achieve target MDA coverage for elimination of lymphatic filariasis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2928210/

Record 1472

pmcid: PMC4617368

pmid: 20799509

doi:

title: Current and future applications of magnetic resonance imaging (MRI) to breast and ovarian cancer patient management.

authors

- Klostergaard J
- Parga K
- Raptis RG

journal: Puerto Rico health sciences journal

journalAbbr: P R Health Sci J

year: 2010

pubdate: 2010 Sep

volume: 29

issue: 3

pages: 223-31

abstract: Magnetic resonance imaging (MRI) is occupying an increasing niche in the clinical diagnostic workup of several cancers, including breast cancers. Despite the high level of implementation of mammography, it has become apparent that MRI can play at least a complementary role in the imaging and diagnosis of primary breast cancers, including ductal carcinoma in situ, the earliest stage of breast cancer that is associated with an increased risk of invasive breast cancer. This can also be said of inflammatory breast cancer, of low incidence but with high impact on overall breast cancer mortality rates, and for which mammography is not ideal due to the typically diffused nature of this disease. Much of the value of breast MRI is dependent on its high sensitivity, resulting from the use of contrast agent enhancement in the detection of breast cancer. Interest has also increased in the application of diffusion-weighted MRI for early assessment of treatment response in this disease. Regarding ovarian and other gynecological cancers, MRI has already demonstrated value in the evaluation of patients with ovarian masses, uterine leiomyoma, endometrioma, and cervical cancer. Features on MRI suggestive of malignant ovarian tumors are varied, and span irregular or solid components to a cystic mass, prominent septations, evidence of peritoneal, hematogenous, or lymphatic spread, or local invasion. The majority of ovarian malignancies are diagnosed in advanced, incurable stages, where exploratory laparotomy provides the opportunity for maximal debulking. Although a role for MRI has yet to be established in this initial setting or in staging, some studies have shown that high sensitivity may be achieved with contrast agent-enhanced MRI for detection of recurrent disease, including demonstration of macroscopic intraabdominal dissemination and the hallmark omental "cake". Efforts in recent years have been focused on design of MRI contrast agents (MRI-CAs), which either target biomarkers, or take advantage of the different physiology of cancerous cells. MRI-CAs based on gadolinium complexes, ferrumoxides, or other metallic nanoparticles have been investigated. This review will focus on the recent progress in the application of MRI to the imaging of breast and ovarian cancers, and present a possible role for molecularly-targeted contrast agents in enriching the context for MRI-based diagnosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4617368/

Record 1473

pmcid: PMC2924526

pmid: 20814491

doi: 10.4103/0972-2327.64628

title: Diagnosis and treatment of chronic insomnia.

authors

- Saddichha S

journal: Annals of Indian Academy of Neurology

journalAbbr: Ann Indian Acad Neurol

year: 2010

pubdate: 2010 Apr

volume: 13

issue: 2

pages: 94-102

abstract: Insomnia is a disorder characterized by inability to sleep or a total lack of sleep, prevalence of which ranges from 10 to 15% among the general population with increased rates seen among older ages, female gender, White population and presence of medical or psychiatric illness. Yet this condition is still under-recognized, under-diagnosed, and under-treated. This article aims to review the operational definitions and management of chronic insomnia. A computerized search on PubMed carried from 1980 to January 2009 led to the summarization of the results. There are several strategies to manage chronic insomnia. To initiate treatment, it is necessary to define it and differentiate it from other co-morbid psychiatric disorders. Non-pharmacologic strategies such as stimulus control therapy and relaxation and cognitive therapies have the best effect sizes followed by sleep restriction, paradoxical intention and sleep hygiene education which have modest to less than modest effect sizes. Among pharmacotherapeutic agents, non-benzodiazepine hypnotics are the first line of management followed by benzodiazepines, amitryptiline and antihistaminics. However, adequate trials of combined behavior therapy and pharmacotherapy are the best course of management.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2924526/

Record 1474

pmcid: PMC3013630

pmid: 20868458

doi: 10.1111/j.1399-5618.2010.00850.x

title: Double-blind randomized trial of risperidone versus divalproex in pediatric bipolar disorder.

authors

- Pavuluri MN
- Henry DB
- Findling RL
- Parnes S
- Carbray JA
- Mohammed T
- Janicak PG
- Sweeney JA

journal: Bipolar disorders

journalAbbr: Bipolar Disord

year: 2010

pubdate: 2010 Sep

volume: 12

issue: 6

pages: 593-605

abstract: OBJECTIVE: To determine the relative effects of risperidone and divalproex in pediatric mania. METHODS: This is a double-blind, randomized, outpatient clinical trial with 66 children and adolescents (mean age= 10.9 ± 3.3 years; age range= 8-18 years) with mania who were randomly assigned to either risperidone (0.5-2 mg/day, n= 33) or divalproex (60-120 µg/mL, n= 33) for a six-week period. Measures included the Young Mania Rating Scale (YMRS) and Child Depression Rating Scale-Revised (CDRS-R). RESULTS: Mixed-effects regression models, with interaction between time and the active drug as predictors, found that the risperidone group had more rapid improvement than the divalproex group ($p < 0.05$), although final scores did not differ significantly between groups. Mixed models using only those subjects who completed the six-week study found similar results. The response rate on YMRS was 78.1% for risperidone and 45.5% for divalproex ($p < 0.01$). The remission rate for risperidone was 62.5%, compared with 33.3% for divalproex ($p < 0.05$). Improvement on the CDRS-R was significantly higher for the risperidone group relative to the divalproex group ($p < 0.05$). There were no significant differences between groups in safety, but subject retention was significantly higher at study endpoint in the risperidone group ($p < 0.01$). Dropout rate was 24% in the risperidone group and 48% in the divalproex group, with increased irritability being the most common reason for dropout in the latter. There was no significant weight gain in either group. CONCLUSION: Results suggest that risperidone was associated with more rapid improvement and greater reduction in manic symptoms compared to divalproex. Although the results suggest that both drugs are safe, risperidone's lower attrition rate and lower rate of adverse events may suggest better toleration. Clinical trials with larger samples are required to confirm these preliminary findings.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3013630/

Record 1475

pmcid: PMC2860825

pmid: 20393092

doi:

title: Drug-induced gynecomastia in children and adolescents.

authors

- Goldman RD

journal: Canadian family physician Medecin de famille canadien

journalAbbr: Can Fam Physician

year: 2010

pubdate: 2010 Apr

volume: 56

issue: 4

pages: 344-5

abstract: QUESTION: I frequently see adolescent boys in my practice with transient gynecomastia. My management includes reassuring the boys and their families; however, I also understand that specific medication, alcohol, and drugs can cause gynecomastia. How common is this phenomenon, and what medications can induce gynecomastia? ANSWER: While gynecomastia is a physiologic phenomenon in most newborns and adolescents, it is important to consider pathologic conditions and medications that can cause breast enlargement. Antibiotics, antiulcer drugs, growth hormones, and chemotherapy have been reported to induce gynecomastia. Adolescents who use anabolic steroids, or who abuse alcohol, marijuana, heroin, or amphetamines, should be alerted to the fact that gynecomastia might develop. Treatment of drug-induced

gynecomastia includes discontinuation of the offending drug. Very rarely is surgical intervention required.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2860825/

Record 1476

pmcid: PMC2913824

pmid: 20697526

doi: 10.3747/co.v17i4.525

title: Ductal carcinoma in situ in a 25-year-old man presenting with apparent unilateral gynecomastia.

authors

- Coroneos CJ
- Hamm C

journal: Current oncology (Toronto, Ont.)

journalAbbr: Curr Oncol

year: 2010

pubdate: 2010 Aug

volume: 17

issue: 4

pages: 133-7

abstract: Ductal carcinoma in situ (DCIS) in a young man is rarely reported. Our patient, a 25-year-old man, presented with apparent symptomatic unilateral gynecomastia. He has a strong history of cancer on both the maternal and paternal sides of his family, including breast and lung (maternal) and melanoma, colon, and pancreatic (paternal). His mother tested negative for BRCA1 and BRCA2. There is no information on paternal genetic testing.The patient was treated with left subcutaneous mastectomy. Upon histologic review of the sample, concurrent gynecomastia and DCIS were discovered. To date, only 4 cases of gynecomastia and DCIS have been described in younger male patients. Because only 30%-50% of patients with DCIS eventually develop invasive cancer in the subsequent 10-20 years, DCIS prevalence in the general population may be higher than predicted. This case underscores the importance of family history in any patient presenting with a breast mass. Patients must be made aware of the risk, however small it may be, and physicians must remain cautious of cancer in young men with gynecomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2913824/

Record 1477

pmcid: PMC2807736

pmid: 19330525

doi: 10.1007/s10549-009-0379-0

title: Etiologic factors for male breast cancer in the U.S. Veterans Affairs medical care system database.

authors

- Brinton LA
- Carreon JD
- Gierach GL
- McGlynn KA
- Gridley G

journal: Breast cancer research and treatment

journalAbbr: Breast Cancer Res Treat

year: 2010

pubdate: 2010 Jan

volume: 119

issue: 1

pages: 185-92

abstract: The etiology of male breast cancer is largely unknown, reflecting its relative rarity. Although a number of previous studies have suggested relationships with a variety of medical conditions, the results have largely derived from case-control studies and may reflect recall biases. Within the large U.S. Veterans Affairs computerized medical care system database, we had the opportunity to access 26 million hospital discharge records over the period 1969-1996 and to relate various documented medical conditions to the risk of subsequent male breast cancer. This allowed us to calculate relative risks (RR) and 95% confidence intervals (CI) for male breast cancer associated with conditions occurring one or more years after initial hospitalization, adjusted for age, race, calendar year, duration of follow-up, and number of hospital visits. Among 4,501,578 men aged 18-100 years, a total of 642 cases of primary male breast cancer were identified (523 among whites, 119 among blacks). Medical conditions that were significantly related to risk were diabetes (RR 1.30, 95% CI 1.05-1.60), obesity (1.98, 1.55-2.54), orchitis/epididymitis (1.84, 1.10-3.08), Klinefelter syndrome (29.64, 12.26-71.68), and gynecomastia (5.86, 3.74-9.17). Additionally, among black patients, cholelithiasis emerged as a significant risk predictor (3.45, 1.59-7.47). Diseases that have previously been related to male breast cancer risk that were not supported by our study results included thyroid diseases, smoking-related conditions, liver cirrhosis, prostatic hyperplasia, and fractures. After adjustment for obesity, the association with diabetes disappeared, but that with gynecomastia persisted. In multivariate models that simultaneously considered all important medical predictors of risk, significant risks were seen for Klinefelter syndrome (16.83, 6.81-41.62), gynecomastia (5.08, 3.21-8.03), obesity (1.91, 1.50-2.44), and orchitis/epididymitis (1.80, 1.08-3.01). These results support previous speculations that male breast cancer is influenced not only by tissue at risk, but also by hormonal and inflammatory factors.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2807736/

Record 1478

pmcid: PMC3075822

pmid: 20552926

doi:

title: Gabapentin for the management of hot flashes in prostate cancer survivors: a longitudinal continuation Study-NCCTG Trial N00CB.

authors

- Moraska AR
- Atherton PJ
- Szydlo DW
- Barton DL
- Stella PJ
- Rowland KM Jr
- Schaefer PL
- Krook J
- Bearden JD
- Loprinzi CL

journal: The journal of supportive oncology

journalAbbr: J Support Oncol

year: 2010

pubdate: 2010 May-Jun

volume: 8

issue: 3

pages: 128-32

abstract: Hot flashes are a complication of androgen deprivation therapy for prostate cancer. A phase III study showed that use of low-dose gabapentin was well tolerated and moderately decreased the frequency of hot flashes due to androgen deprivation therapy when taken for 4 weeks. The current study, an open-label continuation of the randomized study, examined the efficacy and toxicity of gabapentin when taken for (an additional) 8 weeks. Patients were allowed to start, or continue, gabapentin and to titrate the dose to maximum efficacy, up to 900 mg/d. They were asked to complete a hot flash diary daily and keep weekly logs of toxicity, satisfaction with hot flash control, and quality of life. The moderate reduction in hot flash frequency and severity in the randomized phase of the study appeared to be maintained during this continuation phase. Men originally receiving the placebo or lowest dose of gabapentin (300 mg/d) had improved hot flash control relative to that at the end of the randomized phase. Minimal adverse effects were reported. These findings suggest that low-dose gabapentin is moderately efficacious for at least 12 weeks of hot flash treatment in men undergoing androgen deprivation therapy for prostate cancer and seems to be well tolerated. (NCT00028572)

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3075822/

Record 1479

pmcid: PMC3082815

pmid: 21526089

doi:

title: Granulosa cell tumor in a six-year-old girl presented as precocious puberty.

authors

- Hashemipour M
- Moaddab MH
- Nazem M
- Mahzouni P
- Salek M

journal: Journal of research in medical sciences : the official journal of Isfahan University of Medical Sciences

journalAbbr: J Res Med Sci

year: 2010

pubdate: 2010 Jul

volume: 15

issue: 4

pages: 240-2

abstract: Ovarian sex cord-stromal tumors, including granulose cell tumors (GCTs), are a group of neoplasms that occur rarely, especially in children. Only 0.1 percent of all ovarian tumors and 4-5 percent of GCTs occur in children. The most common presentation of these tumors in children is precocious puberty. We report a 6 years old girl with isosexual precocity, presented as premature thelarche and vaginal bleeding. Ultrasonography of pelvis showed a hypoechoic solid mass of left ovary.Microscopic features of the resected mass were characteristic of juvenile GCT. Although in most of girls with precocious puberty, the etiology is idiopathic, important causes, such as ovarian tumors must be considered.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3082815/

Record 1480

pmcid: PMC3303362

pmid: 22470745

doi: 10.3941/jrcr.v4i7.452

title: Gynaecomastia associated with highly active antiretroviral therapy (HAART).

authors

- Meerkotter D

journal: Journal of radiology case reports

journalAbbr: J Radiol Case Rep

year: 2010

pubdate: 2010

volume: 4

issue: 7

pages: 34-40

abstract: There is a global Acquired Immune Deficiency Syndrome (AIDS) epidemic. It is estimated that the total number of people living with Human Immune deficiency Virus (HIV) is 33.2 million in the year 2007. An estimated 2.1 million people died from AIDS in 2007. As highly active antiretroviral therapy (HAART) becomes more widely used, the side effects of this regime are becoming more evident in clinical practice. We present 2 cases that demonstrate the association of HAART and gynaecomastia. Imaging features of gynaecomastia and the differential for these changes will be reviewed. Proposed mechanisms for the pathogenesis of gynaecomastia will be discussed. Possible therapeutic options will also be briefly considered.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3303362/

Record 1481

pmcid: PMC2947973

pmid: 20884830

doi: 10.4065/mcp.2010.0093

title: Gynecomastia.

authors

- Nuttall FQ

journal: Mayo Clinic proceedings

journalAbbr: Mayo Clin Proc

year: 2010

pubdate: 2010 Oct

volume: 85

issue: 10

pages: 961-2

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2947973/

Record 1482

pmcid: PMC2837521

pmid: 20195108

doi: 10.1097/PRS.0b013e3181cb5e29

title: Health characteristics of postmenopausal women with breast implants.

authors

- Rubin JP
- Landfair AS
- Shestak K
- Lane D
- Valoski A
- Chang Y
- Tindle HA
- Kuller LH

journal: Plastic and reconstructive surgery

journalAbbr: Plast Reconstr Surg

year: 2010

pubdate: 2010 Mar

volume: 125

issue: 3

pages: 799-810

abstract: BACKGROUND: Long-term health characteristics and quality of life in patients with breast implants are important issues in plastic surgery. METHODS: The authors evaluated characteristics of women who had breast implant surgery in the Women's Health Initiative observational study between 1993 and 1998. Most women in this study cohort had breast implant surgery 20 or more years before recruitment into the study. The women who were in the study who had not undergone breast implant surgery served as the comparison group. There were 86,686 women in the study who did not have breast implant surgery and an absent history of breast cancer, and 1257 women who had breast implant surgery and no prior breast cancer. RESULTS: Total mortality rates were substantially lower among women with breast implants, as was the incidence of coronary heart disease. Women with breast implants in this study had a lower body mass index throughout adult life and were more physically active than control subjects. After adjustment for these variables, differences in total mortality were no longer statistically significant. Women who had breast implants reported overall poorer quality of life and emotional well-being. These differences were small, but statistically significant. Among women with breast implant surgery, 7 percent of deaths were due to suicide (n = 3) versus 0.4 percent (n = 20) in controls. CONCLUSIONS: Significant differences in health characteristics and quality-of-life measures are seen in a cohort of women with breast implants decades after implant surgery. Further longitudinal studies need to focus on both physical and psychological health among women undergoing breast implant surgery.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2837521/

Record 1483

pmcid: PMC2943074

pmid: 20871665

doi: 10.1155/2010/159402

title: Hyperprolactinemia with antipsychotic drugs in children and adolescents.

authors

- Rosenbloom AL

journal: International journal of pediatric endocrinology

journalAbbr: Int J Pediatr Endocrinol

year: 2010

pubdate: 2010

volume: 2010

issue:

pages:

abstract: There is increasing use of antipsychotic drugs in pediatric and psychiatry practice for a wide range of behavioral and affective disorders. These drugs have prominent side effects of interest to pediatric endocrinologists, including weight gain and associated metabolic risk factors and hyperprolactinemia. The drugs block dopamine action, thus disinhibiting prolactin secretion. Hyperprolactinemia is especially prominent with first-generation antipsychotics such as haloperidol and the second-generation drugs, most commonly risperidone, with some patients developing gynecomastia or galactorrhea or, as a result of prolactin inhibition of gonadotropin releasing hormone from the hypothalamus, amenorrhea. With concern about the long-term effects of antipsychotics on bone mass and pituitary tumor formation, it is prudent to monitor serum prolactin levels in antipsychotic drug-treated pediatric patients and consider treatment with an agent less likely to induce hyperprolactinemia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2943074/

Record 1484

pmcid: PMC3093996

pmid: 21589832

doi: 10.4081/pr.2010.e19

title: Impact of child obesity on adipose tissue physiology: assessment of adipocytokines and inflammatory cytokines as biomarkers of obesity.

authors

- Leoni MC
- Valsecchi C
- Mantelli M
- Marastoni L
- Tinelli C
- Marchi A
- Castellazzi A

journal: Pediatric reports

journalAbbr: Pediatr Rep

year: 2010

pubdate: 2010 Sep 6

volume: 2

issue: 2

pages: e19

abstract: Obesity could be interpreted as a low grade inflammatory state. The role of cytokines for innate and acquired immune response and adipocytokines in pathogenesis of obesity is not completely understood. The aim of the study was to evaluate anthropometric parameters, adipocytokines and inflammatory cytokine levels as biomarkers of childhood obesity. This investigation was designed as a longitudinal observational study. Forty-seven obese children (19 males and 28 females) were enrolled by Pediatric Clinic of the Foundation IRCCS Policlinico San Matteo, Pavia, Italy. For each patients a blood sample, used for other biochemical evaluations,

was collected. Cytokines and adipocytokines plasmatic levels were determined using an ELISA method. Plasma leptin levels are in correlation with age (r=0.5; P<0.001) and BMI-z score (r=0.36; P<0.001), particularly in girls; plasma resistin levels are in inverse correlation with age, particularly in boys (r=-0.67; P<0.001) and in correlation with BMI-z score (r=0.52; P=0.002). Plasma leptin and resistin levels show a good correlation with antrophometric parameters of child obesity (sex and BMI z score). This study suggests that leptin and resistin can be considered as biomarker of childhood obesity and its comorbility. We observed a statistically significant correlation between plasma leptin and resistin levels and antrophometric parameters of child obesity (sex and BMI z score). This study suggests that adipocytokines, such as leptin and resistin, can be considered as biomarkers of childhood obesity.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3093996/

Record 1485

pmcid: PMC2928758

pmid: 20707918

doi: 10.1186/1471-230X-10-92

title: Infection with HIV and HCV enhances the release of fatty acid synthase into circulation: evidence for a novel indicator of viral infection.

authors

- Aragonès G
- Alonso-Villaverde C
- Oliveras-Ferraros C
- Beltrán-Debón R
- Rull A
- Rodríguez-Sanabria F
- Camps J
- Martín AV
- Menéndez JA
- Joven J

journal: BMC gastroenterology

journalAbbr: BMC Gastroenterol

year: 2010

pubdate: 2010 Aug 13

volume: 10

issue:

pages: 92

abstract: BACKGROUND: Fatty acid synthase (FASN) is an enzyme synthesized by the liver and plays an important role in lipogenesis. The present study aimed to investigate whether serum FASN concentration may provide a direct link between HIV and/or HCV viral infections and lipid metabolic disorders commonly observed in HIV/HCV-infected patients. METHODS: We evaluated serum FASN concentration in 191 consecutive HIV-infected patients in the absence or presence of HCV co-infection. For comparison, 102 uninfected controls were included. Metabolic and inflammatory phenotype was also compared with respect to the presence of HCV co-infection. RESULTS: Serum FASN concentration was significantly higher in HIV-infected patients than in healthy participants and HCV co-infected patients showed higher levels than those without co-infection. Levels were also affected by treatment regimen, but marginally influenced by virological variables. Insulin concentration was the sole variable among metabolic parameters that demonstrated a significant correlation with serum FASN concentrations. Serum alanine aminotransferase (ALT) values correlated significantly with serum FASN

concentration and provided the best discrimination with respect to the presence or absence of HCV co-infection. In multivariate analysis, only ALT, monocyte chemoattractant protein-1 (MCP-1) and the presence of antiretroviral treatment regimen significantly contributed to explain serum FASN concentration in HIV/HCV co-infected patients. CONCLUSION: Serum FASN concentration is significantly increased in HIV-infected individuals. The release of FASN into the circulation is further enhanced in patients who are co-infected with HCV. Subsequent studies should explore the usefulness of this indicator to monitor the effect of viral infections on disease progression and survival.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2928758/

Record 1486

pmcid: PMC2987341

pmid: 20145678

doi: 10.1038/ejhg.2009.236

title: Mammary-digital-nail (MDN) syndrome: a novel phenotype maps to human chromosome 22q12.3-13.1.

authors

- Genzer-Nir M
- Khayat M
- Kogan L
- Cohen HI
- Hershkowitz M
- Geiger D
- Falik-Zaccai TC

journal: European journal of human genetics : EJHG

journalAbbr: Eur J Hum Genet

year: 2010

pubdate: 2010 Jun

volume: 18

issue: 6

pages: 662-7

abstract: Mammary-digital-nail syndrome is a novel phenotypic association consisting of anonychia onychodystrophy with hypoplasia or absence of distal phalanges in males and females, accompanied by juvenile hypertrophy of the breast in affected females. This newly described genetic trait presents an autosomal dominant inheritance pattern, with either reduced penetrance or germ-line mosaicism. Analysis of the pedigree, linkage studies followed by a genome-wide screen and by haplotype analysis defined the locus for the phenotype within a 12 cM (4.3 Mb) interval on chromosome 22q12.3-13.1. This chromosomal region has not been implicated before in genetic disorders of the mammary tissue or limbs. These data suggest a possibly novel signaling pathway affecting the organogenesis of limbs and mammary glands in humans.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2987341/

Record 1487

pmcid: PMC3080063

pmid: 20412670

doi: 10.1308/003588410X12628812458815

title: Minimally invasive excision of gynaecomastia--a novel and effective surgical technique.

authors

- Qutob O
- Elahi B
- Garimella V
- Ihsan N
- Drew PJ

journal: Annals of the Royal College of Surgeons of England

journalAbbr: Ann R Coll Surg Engl

year: 2010

pubdate: 2010 Apr

volume: 92

issue: 3

pages: 198-200

abstract: INTRODUCTION: More aesthetically acceptable treatment options have been sought to minimise the morbidity associated with open surgery for gynaecomastia. This study investigated the use of a vacuum-assisted biopsy device (VABD) and liposuction to provide minimally invasive approach. PATIENTS AND METHODS: Patients diagnosed with idiopathic benign gynaecomastia referred to the Breast Care Unit of Castle Hill Hospital between June 2002 and April 2007 and requesting surgical intervention underwent VABD excision and liposuction. All patients underwent thorough investigations to exclude any underlying cause for their gynaecomastia. The procedure was carried out by a single consultant surgeon with special interest in breast surgery. An 8-G mammotome probe was advanced through a 4-mm incision positioned in the corresponding anterior axillary line to excise the glandular disc. Liposuction was performed through the same incision. Incision wounds were closed with Steristrips. A pressure dressing was applied over wound by corset and an inflatable device. RESULTS: Thirty-six male patients with grade I and II gynaecomastia were recruited (22 bilateral, 14 unilateral). Average age was 33.3 years (range, 16-88 years). All underwent mammotome excision and liposuction. There were no conversions to an open procedure. The average procedure time was 50.3 min (range, 30-80 min). One intra-operative complication was recorded. The minimum follow-up time was 2 months. Thirty-four patients reported excellent satisfaction, two patients had residual gynaecomastia and needed a re-do procedure. Three patients developed small haematomas that resolved spontaneously. CONCLUSION: This novel, minimally invasive, surgical approach for gynaecomastia gives excellent results with minimal morbidity.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3080063/

Record 1488

pmcid: PMC3218685

pmid: 22096395

doi: 10.2147/HIV.S13429

title: New and emerging agents in the management of lipodystrophy in HIV-infected patients.

authors

- Bonnet E

journal: HIV/AIDS (Auckland, N.Z.)

journalAbbr: HIV AIDS (Auckl)

year: 2010

pubdate: 2010

volume: 2

issue:

pages: 167-78

abstract: Lipodystrophy remains a major long-term complication in human immunodeficiency virus-infected patients under antiretroviral (ARV) therapy. Patients may present with lipoatrophy or lipohypertrophy or both. The choice of treatments to improve fat redistribution depends on the form of lipodystrophy and its duration. Measures known to improve lipoatrophy are switches in ARV therapy (stavudine or zidovudine to abacavir or tenofovir) and filling interventions. Pioglitazone may be added to these measures, although any benefits appear small. Uridine and leptin were found to be disappointing so far. Regarding lipohypertrophy, diet and exercise, recombinant human growth hormone, and metformin may reduce visceral fat, but may worsen subcutaneous lipoatrophy. Surgical therapy may be required. Attractive pharmacologic treatments include growth hormone-releasing factor and leptin. Adiponectin and adiponectin receptors are promising therapeutic targets to explore.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3218685/

Record 1489

pmcid: PMC2864863

pmid: 20461190

doi: 10.3348/kjr.2010.11.3.355

title: Rapidly growing bilateral pseudoangiomatous stromal hyperplasia of the breast.

authors

- Ryu EM
- Whang IY
- Chang ED

journal: Korean journal of radiology

journalAbbr: Korean J Radiol

year: 2010

pubdate: 2010 May-Jun

volume: 11

issue: 3

pages: 355-8

abstract: A tumoral pseudoangiomatous stromal hyperplasia (PASH) that causes huge breast enlargement is very rare. Only two cases of huge tumoral PASHs have been reported in the English medical literature. We report here on a surgically confirmed case of bilateral huge tumoral PASH in a 47-year-old woman, and we present the imaging and histopathology findings. We also review the relevant medical literature.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2864863/

Record 1490

pmcid: PMC3125058

pmid: 21731261

doi: 10.4103/0973-029X.72499

title: Review of paraneoplastic syndromes associated with oropharyngeal squamous cell carcinoma.

authors

- Mathew DG
- Rooban T
- Janani V
- Joshua E

- Rao U
- Ranganathan K

journal: Journal of oral and maxillofacial pathology : JOMFP

journalAbbr: J Oral Maxillofac Pathol

year: 2010

pubdate: 2010 Jul

volume: 14

issue: 2

pages: 41-7

abstract: Malignancies are usually preceded by the presence of various paraneoplastic syndromes (PNS), which could be the indirect and/or remote effects of the metabolites produced by neoplastic cells. PNS manifested by oropharyngeal squamous cell carcinomas, which is the most common head and neck malignancy, are highlighted in this review. Knowledge of the clinical spectrum of these syndromes will equip the oral physician for early diagnosis and management of these hidden malignancies, especially of the pharyngeal region.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3125058/

Record 1491

pmcid: PMC2933084

pmid: 20806366

doi: 10.1002/pbc.22574

title: T-cell acute lymphoblastic leukemia in association with Börjeson-Forssman-Lehmann syndrome due to a mutation in PHF6.

authors

- Chao MM
- Todd MA
- Kontny U
- Neas K
- Sullivan MJ
- Hunter AG
- Picketts DJ
- Kratz CP

journal: Pediatric blood & cancer

journalAbbr: Pediatr Blood Cancer

year: 2010

pubdate: 2010 Oct

volume: 55

issue: 4

pages: 722-4

abstract: Börjeson-Forssman-Lehmann syndrome (BFLS) is a rare X-linked mental retardation syndrome that is caused by germline mutations in PHF6. We describe a 9-year-old male with BFLS, who developed T-cell acute lymphoblastic leukemia (T-ALL). The PHF6 gene is located on the X chromosome and encodes a protein with two PHD-type zinc finger domains and four nuclear localization sequences. Previously, overexpression of Phf6 was observed in murine T-cell lymphomas. Our observation indicates that BFLS may represent a cancer

predisposition syndrome and that mutations of PHF6 contribute to T-ALL.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2933084/

Record 1492

pmcid: PMC2963653

pmid: 20863580

doi: 10.1016/j.tips.2010.08.005

title: Therapeutic approaches to spinal and bulbar muscular atrophy.

authors

- Ranganathan S
- Fischbeck KH

journal: Trends in pharmacological sciences

journalAbbr: Trends Pharmacol Sci

year: 2010

pubdate: 2010 Nov

volume: 31

issue: 11

pages: 523-7

abstract: Spinal and bulbar muscular atrophy is a hereditary motor neuron disease caused by trinucleotide repeat expansion in the androgen receptor gene. The disease mechanism probably involves a toxic gain of function in the mutant protein, because other mutations that cause a loss of androgen receptor function result in a different phenotype and the mutant protein is toxic in mouse models. In these models, the toxicity is ligand-dependent and is associated with protein aggregation, as well as altered transcriptional regulation, axonal transport and mitochondrial function. Various therapeutic approaches have shown efficacy in mouse models, including androgen reduction, heat shock protein 90 (HSP90) inhibition and insulin-like growth factor (IGF)-1 overexpression. Clinical trials of androgen-reducing agents have had mixed results, with indications of efficacy but no proof of clinically meaningful benefit to date. These clinical studies have established outcome measures for future trials of other agents that have been beneficial in animal studies.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2963653/

Record 1493

pmcid: PMC5649796

pmid: 29147205

doi: 10.4021/wjon245w

title: Twelve Years of Experience in the Management of Testicular Germ Cell Tumors at a Referral Center in Portugal.

authors

- Valadares D
- Nery F
- Marques F

journal: World journal of oncology

journalAbbr: World J Oncol

year: 2010

pubdate: 2010 Oct

volume: 1

issue: 5

pages: 187-193

abstract: BACKGROUND: Testicular germ cell tumors (TGCT) are generally rare but quite frequent in young males. Guidelines are well established for their management. METHODS: We present the first report from Portugal on clinical, histological, treatment modalities and outcomes of a population with TGCT. Data was retrospectively analyzed for the 1996 through 2008 period, applying a previous internally validated protocol. RESULTS: Seventy nine patients with TGCT were identified, 40.5% had seminomatous and 59.5% nonseminomatous tumors. Incidence rates were higher among males in their twenties and thirties. Pain and swelling testis were the most common symptoms and microlithiasis was detected in 20.3% of patients. Lower stages were more frequent in seminomatous tumors. Orchiectomy was done in all patients and further therapy was performed by guidelines recommendations in 86.1% of them. Hematological toxicity was found in 44.3% of the population studied and free disease survival rates were at 88.6%. CONCLUSIONS: This retrospective study corroborates the European Western country trends concerning TGCT. Mortality was only seen in nonseminomatous TGCT group. Good risk and lower TGCT stages have no deaths reported. Public health campaigns should be undertaken to guide patients to seek medical advice earlier in the course of the disease.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5649796/

Record 1494

pmcid: PMC3827596

pmid: 24281037

doi: 10.3390/cancers2010143

title: What is inflammatory breast cancer? Revisiting the case definition.

authors

- Levine PH
- Zolfaghari L
- Young H
- Hafi M
- Cannon T
- Ganesan C
- Veneroso C
- Brem R
- Sherman M

journal: Cancers

journalAbbr: Cancers (Basel)

year: 2010

pubdate: 2010 Mar 3

volume: 2

issue: 1

pages: 143-52

abstract: The case definition for inflammatory breast cancer (IBC) is controversial. The American Joint Committee on Cancer defines IBC as redness, warmth and edema involving at least half the breast. The SEER program relies on a pathologic finding of dermal lymphatic invasion and recently added those with clinical involvement of more than 3/4 of the breast. We established a registry to collect information and specimens from

IBC patients to clarify the epidemiology and biology of these tumors. The goals of this report are to suggest improvements regarding case definitions and provide data on the variety of presentations relevant to early diagnosis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3827596/

Record 1495

pmcid: PMC2761812

pmid: 19829767

doi:

title: 2009 AUA-NS Abstracts.

authors

journal: Canadian Urological Association journal = Journal de l'Association des urologues du Canada

journalAbbr: Can Urol Assoc J

year: 2009

pubdate: 2009 Oct

volume: 3

issue: 5 Suppl 3

pages: S132-76

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2761812/

Record 1496

pmcid: PMC3600416

pmid: 19593179

doi: 10.1097/JCP.0b013e3181ac935c

title: A randomized, double-blind, placebo-controlled trial of oral Matricaria recutita (chamomile) extract therapy for generalized anxiety disorder.

authors

- Amsterdam JD
- Li Y
- Soeller I
- Rockwell K
- Mao JJ
- Shults J

journal: Journal of clinical psychopharmacology

journalAbbr: J Clin Psychopharmacol

year: 2009

pubdate: 2009 Aug

volume: 29

issue: 4

pages: 378-82

abstract: OBJECTIVE: We conducted a randomized, double-blind, placebo-controlled efficacy and tolerability trial of Matricaria recutita (chamomile) extract therapy in patients with mild to moderate generalized anxiety disorder (GAD). We hypothesized that chamomile would be superior to placebo in reducing GAD symptoms with a comparable tolerability profile. MATERIALS AND METHODS: Sixty-one outpatients with mild to moderate GAD were enrolled, and 57 were randomized to either double-blind chamomile extract (n = 28) or placebo therapy (n = 29) for 8 weeks. The study was powered to detect a statistically significant and clinically meaningful group difference in change over time in total Hamilton Anxiety Rating (HAM-A) scores. Secondary outcomes included change in the Beck Anxiety Inventory, Psychological Well Being, and Clinical Global Impression Severity scores and the proportion of patients with 50% reduction or more in baseline HAM-A score. RESULTS: We observed a significantly greater reduction in mean total HAM-A score during chamomile versus placebo therapy (P = 0.047). Although the study was not powered to identify small to moderate differences in secondary outcomes, we observed a positive change in all secondary outcomes in the same direction as the primary outcome measure. One patient in each treatment group discontinued therapy for adverse events. The proportion of patients experiencing 0, 1, 2, or 3 adverse events or more was not significantly different between groups (P = 0.417). CONCLUSIONS: This is the first controlled clinical trial of chamomile extract for GAD. The results suggest that chamomile may have modest anxiolytic activity in patients with mild to moderate GAD. Future studies are needed to replicate these observations.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3600416/

Record 1497

pmcid: PMC2986486

pmid: 19156171

doi: 10.1038/ejhg.2008.261

title: An 8.9 Mb 19p13 duplication associated with precocious puberty and a sporadic 3.9 Mb 2q23.3q24.1 deletion containing NR4A2 in mentally retarded members of a family with an intrachromosomal 19p-into-19q between-arm insertion.

authors

- Lybaek H
- Ørstavik KH
- Prescott T
- Hovland R
- Breilid H
- Stansberg C
- Steen VM
- Houge G

journal: European journal of human genetics : EJHG

journalAbbr: Eur J Hum Genet

year: 2009

pubdate: 2009 Jul

volume: 17

issue: 7

pages: 904-10

abstract: In a 2 and a half-year-old girl with onset of puberty before the age of 5 months, short stature, hand anomalies and severe mental retardation, an 8.9 Mb interstitial 19p13 duplication containing 215 predicted genes was detected. It was initially assumed that the duplication involved the kisspeptin receptor gene, GPR54, known to stimulate induction of puberty, but more refined duplication mapping excluded this possibility. In an attempt to further understand the genotype-phenotype correlation, global gene expression was measured in skin

fibroblasts. The overall expression pattern was quite similar to controls, and only about 25% of the duplicated genes had an expression level that was increased by more than 1.3-fold, with no obvious changes that could explain the precocious puberty. The proband's mother carried a balanced between-arm insertion of the duplicated segment that resembled a pericentric inversion. The same insertion was found in several other family members, including one who had lost a daughter with severe mental retardation and menarche at the age of 10 years. Another close relative was severely mentally retarded, but neither dysmorphic nor microcephalic. His phenotype was initially ascribed to a presumed cryptic chromosome 19 imbalance caused by the 19p-into19q insertion, but subsequent array-CGH detected a 3.9-Mb deletion of 2q23.3q24.1. This novel microdeletion involves seven genes, of which FMNL2, a suggested regulator of Rho-GTPases, and NR4A2, an essential gene for differentiation of dopaminergic neurons, may be critical genes for the proposed 2q23q24 microdeletion syndrome.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2986486/

Record 1498

pmcid: PMC2920692

pmid: 20948762

doi: 10.3410/M1-2

title: Androgen deprivation therapy for prostate cancer: indications, contraindications and possible consequences.

authors

- Karl A
- Konety B

journal: F1000 medicine reports

journalAbbr: F1000 Med Rep

year: 2009

pubdate: 2009 Jan 21

volume: 1

issue:

pages:

abstract: Androgen deprivation therapy (ADT) has been the mainstay of palliative treatment in advanced and metastatic prostate cancer for many years. It is also increasingly being used in patients with localized disease. Here we review some recent trials that are providing information on its benefits and disadvantages in different circumstances.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2920692/

Record 1499

pmcid: PMC7149689

pmid:

doi: 10.1016/B978-012373944-5.00178-4

title: Antiviral Agents.

authors

- Paintsil E
- Cheng YC

journal: Encyclopedia of Microbiology

journalAbbr: Encyclopedia of Microbiology

year: 2009

pubdate: 2009

volume:

issue:

pages: 223-57

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7149689/

Record 1500

pmcid: PMC2628841

pmid: 19155362

doi:

title: Approach to inflammatory breast cancer.

authors

- Molckovsky A
- Fitzgerald B
- Freedman O
- Heisey R
- Clemons M

journal: Canadian family physician Medecin de famille canadien

journalAbbr: Can Fam Physician

year: 2009

pubdate: 2009 Jan

volume: 55

issue: 1

pages: 25-31

abstract: OBJECTIVE: To review the definition, clinical presentation, and management of inflammatory breast cancer in primary care. SOURCES OF INFORMATION: Relevant research and review articles, as well as personal experience of the authors practising in a specialized locally advanced breast cancer program at a comprehensive cancer centre. Evidence is levels II and III. MAIN MESSAGE: Inflammatory breast cancer is a rare disease that typically presents with a rapidly enlarging erythematous breast, often with no discernable breast mass. Identification of warning signs and recognition of clinical symptoms are crucial to prompt diagnosis and appropriate referral. Management in the primary care setting includes treatment of symptoms, psychosocial support, regular surveillance and follow-up, as well as palliative care. CONCLUSION: Family physicians are usually the entry point to the health care system and are well positioned to assess inflammation of the breast and recognize the warning signs of an underlying inflammatory breast cancer. They are also important members of the team that provides support for breast cancer patients and their families during treatment, follow-up, and end-of-life care.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2628841/

Record 1501

pmcid: PMC3036427

pmid: 21326501

doi: 10.1055/s-0029-1222454

title: Central venous obstruction management.

authors

- Kundu S

journal: Seminars in interventional radiology

journalAbbr: Semin Intervent Radiol

year: 2009

pubdate: 2009 Jun

volume: 26

issue: 2

pages: 115-21

abstract: A major challenge in the management of hemodialysis patients is central venous stenosis and obstruction. Placement of central venous catheters has been shown to result in a high incidence of central venous stenosis or obstruction. There has been extensive literature on the treatment of this important and prevalent problem. Treatment options include percutaneous balloon angioplasty and bare metal stents. Unfortunately, all the available treatment options have variable rates of patency, requiring repeated intervention. More recently, covered stents have been mentioned in the literature for the treatment of central venous stenosis and obstruction. There is very little data to date, and further randomized controlled trials will be needed to compare the efficacy of percutaneous balloon angioplasty, bare metal stents, and covered stents. It appears prevention of this difficult problem is paramount, by limiting use of central venous catheters.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3036427/

Record 1502

pmcid: PMC2792370

pmid: 19846582

doi: 10.1093/brain/awp258

title: Clinical features of spinal and bulbar muscular atrophy.

authors

- Rhodes LE
- Freeman BK
- Auh S
- Kokkinis AD
- La Pean A
- Chen C
- Lehky TJ
- Shrader JA
- Levy EW
- Harris-Love M
- Di Prospero NA
- Fischbeck KH

journal: Brain : a journal of neurology

journalAbbr: Brain

year: 2009

pubdate: 2009 Dec

volume: 132

issue: Pt 12

pages: 3242-51

abstract: Spinal and bulbar muscular atrophy is an X-linked motor neuron disease caused by a CAG repeat expansion in the androgen receptor gene. To characterize the natural history and define outcome measures for clinical trials, we assessed the clinical history, laboratory findings and muscle strength and function in 57 patients with genetically confirmed disease. We also administered self-assessment questionnaires for activities of daily living, quality of life and erectile function. We found an average delay of over 5 years from onset of weakness to diagnosis. Muscle strength and function correlated directly with serum testosterone levels and inversely with CAG repeat length, age and duration of weakness. Motor unit number estimation was decreased by about half compared to healthy controls. Sensory nerve action potentials were reduced in nearly all subjects. Quantitative muscle assessment and timed 2 min walk may be useful as meaningful indicators of disease status. The direct correlation of testosterone levels with muscle strength indicates that androgens may have a positive effect on muscle function in spinal and bulbar muscular atrophy patients, in addition to the toxic effects described in animal models.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2792370/

Record 1503

pmcid: PMC2929552

pmid: 20805972

doi: 10.4103/0974-7753.51930

title: Finasteride induced Gynecomastia: Case report and Review of the Literature.

authors

- Ramot Y
- Czarnowicki T
- Zlotogorski A

journal: International journal of trichology

journalAbbr: Int J Trichology

year: 2009

pubdate: 2009 Jan

volume: 1

issue: 1

pages: 27-9

abstract: Finasteride (1 mg/day) is widely utilized by dermatologists for the treatment of androgenetic alopecia. Although enjoying a relatively good safety profile, several sex-related adverse effects have been reported with this drug. Here we report two cases of gynecomastia, one of them bilateral, caused by Propecia((R)) prescribed for the treatment of androgenetic alopecia. Although relatively rare, physicians should be aware of this side effect and inform their patients when prescribing this medication.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2929552/

Record 1504

pmcid: PMC2783854

pmid: 19654551

doi: 10.1097/QAI.0b013e3181b628be

title: Gender differences in discontinuation of antiretroviral treatment regimens.

authors

- Kempf MC
- Pisu M
- Dumcheva A
- Westfall AO
- Kilby JM
- Saag MS

journal: Journal of acquired immune deficiency syndromes (1999)

journalAbbr: J Acquir Immune Defic Syndr

year: 2009

pubdate: 2009 Nov 1

volume: 52

issue: 3

pages: 336-41

abstract: BACKGROUND: Reported reasons for change or discontinuation of antiretroviral therapy ([delta]ART) include adverse events, intolerability, and nonadherence. Little is known how reasons for [delta]ART differ by gender. METHODS: In a retrospective cohort study, rates and reasons for [delta]ART alterations in a large University-based HIV clinic cohort were evaluated. Logistic regression analyses were used to evaluate the relationship between reasons for [delta]ART and gender. Cox proportional hazard models were used to investigate time to [delta]ART. RESULTS: In total, 631 HIV-positive individuals were analyzed. Women (n = 164) and men (n = 467) were equally likely (53.0% and 54.4%, respectively) to discontinue treatment within 12 month of initiating a new regimen. Reasons for [delta]ART, however, were different based on gender--women were more likely to [delta]ART due to poor adherence [adjusted odds ratio (OR), 1.44; 95% confidence interval (CI): 0.85 to 2.42], dermatologic symptoms (adjusted OR, 2.88; 95% CI: 1.01 to 8.18), neurological reasons (adjusted OR, 1.82; 95% CI: 0.98 to 3.39), constitutional symptoms (adjusted OR, 2.23; 95% CI: 1.10 to 4.51), and concurrent medical conditions (adjusted OR, 2.03; 95% CI: 1.00 to 4.12). CONCLUSIONS: Although the rates of [delta]ART are similar among men and women in clinical practice, the reasons for treatment changes are different based on gender. The potential for unique patterns of adverse events and poor adherence among women requires further investigation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2783854/

Record 1505

pmcid: PMC3066743

pmid: 21483532

doi: 10.4176/090428

title: Gender Re-assignment at 17 Medical, Ethical, Religious and Cultural Dilemma!

authors

- Bukar M
- Kwaghe G
- Bakari A
- Audu B
- Ojo E
- Ngilari U

journal: The Libyan journal of medicine

journalAbbr: Libyan J Med

year: 2009

pubdate: 2009 Dec 1

volume: 4

issue: 4

pages: 131-2

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3066743/

Record 1506

pmcid: PMC3027370

pmid: 21686430

doi: 10.1136/bcr.11.2008.1177

title: Gestational gigantomastia.

authors

- John MK
- Rangwala TH

journal: BMJ case reports

journalAbbr: BMJ Case Rep

year: 2009

pubdate: 2009

volume: 2009

issue:

pages:

abstract: A case of massive hypertrophy of the breasts in pregnancy was seen in our institution, which is a tertiary referral centre for the United Arab Emirates region with a delivery rate of 7000/year. It is a very rare condition (1 in 100000) and the only case seen in our hospital over the past 20 years. No similar case has been reported from the United Arab Emirates or Gulf regions, to our knowledge. The patient presented at a gestational age of 18 weeks on account of progressive swelling of the breasts which started at 14 weeks' gestation. In pregnancy she was managed conservatively with analgesics, bromocriptine and breast support. She had bilateral reduction mammoplasty 1 year after delivery. The outcome was satisfactory, and the patient was pleased with the cosmetic result.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3027370/

Record 1507

pmcid: PMC2770912

pmid: 19880691

doi: 10.1016/S0025-6196(11)60671-X

title: Gynecomastia: pathophysiology, evaluation, and management.

authors

- Johnson RE
- Murad MH

journal: Mayo Clinic proceedings

journalAbbr: Mayo Clin Proc

year: 2009

pubdate: 2009 Nov

volume: 84

issue: 11

pages: 1010-5

abstract: Gynecomastia, defined as benign proliferation of male breast glandular tissue, is usually caused by increased estrogen activity, decreased testosterone activity, or the use of numerous medications. Although a fairly common presentation in the primary care setting and mostly of benign etiology, it can cause patients considerable anxiety. The initial step is to rule out pseudogynecomastia by careful history taking and physical examination. A stepwise approach that includes imaging and laboratory testing to exclude neoplasms and endocrinopathies may facilitate cost-effective diagnosis. If results of all studies are normal, idiopathic gynecomastia is diagnosed. The evidence in this area is mainly of observational nature and lower quality.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2770912/

Record 1508

pmcid: PMC2632162

pmid: 19221583

doi:

title: Gynecomastia: what the surgeon needs to know.

authors

- Singer-Granick CJ
- Granick MS

journal: Eplasty

journalAbbr: Eplasty

year: 2009

pubdate: 2009

volume: 9

issue:

pages: e6

abstract: OBJECTIVE: The purpose of this review is to present the complex underlying pathophysiology that can form the basis of this common condition. METHODS: More than 20 years of clinical experience in endocrinology and plastic surgery and a review of the English-language literature were used to form the basis of this review. RESULTS: While idiopathic and physiologic causes are frequent, there are multiple, significant, underlying disorders that can result in gynecomastia, including chronic illness, cancer, medications, syndromes, and a variety of endocrinopathies. CONCLUSION: Both history and physical examination are frequently sufficient to make an appropriate diagnosis. In patients who do not have a definitive etiology of their gynecomastia, a screening battery of laboratory tests is sufficient to rule out significant pathophysiology, although these tests may be difficult to interpret in children and adolescents. An endocrinology consultation is suggested whenever an abnormal screening laboratory test occurs or if there are any other suggestions of underlying endocrinopathy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2632162/

Record 1509

pmcid: PMC2919754

pmid: 20706598

doi: 10.4102/sajhivmed.v10i4.264

title: LIPODYSTROPHY SYNDROME IN HIV-INFECTED CHILDREN ON HAART.

authors

- Innes S
- Levin L
- Cotton M

journal: Southern African journal of HIV medicine

journalAbbr: South Afr J HIV Med

year: 2009

pubdate: 2009 Dec

volume: 10

issue: 4

pages: 76-80

abstract: Lipodystrophy syndrome (LD) is common in HIV-infected children, particularly those taking didanosine, stavudine or zidovudine. Lipo-atrophy in particular causes major stigmatisation and interferes with adherence. In addition, LD may have significant long-term health consequences, particularly cardiovascular. Since the stigmatising fat distribution changes of LD are largely permanent, the focus of management remains on early detection and arresting progression. Practical guidelines for surveillance and avoidance of LD in routine clinical practice are presented. The diagnosis of LD is described and therapeutic options are reviewed. The most important therapeutic intervention is to switch the most likely offending antiretroviral to a non-LD-inducing agent as soon as LD is recognised. Typically, when lipoatrophy or lipohypertrophy is diagnosed the thymidine nucleoside reverse transcriptase inhibitor (NRTI) is switched to a non-thymidine agent such as abacavir (or tenofovir in adults). Where dyslipidaemia is predominant, a dietician review is helpful, and the clinician may consider switching to a protease inhibitor-sparing regimen or to atazanavir.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2919754/

Record 1510

pmcid: PMC3074615

pmid: 18952456

doi: 10.1016/j.critrevonc.2008.09.004

title: Management of complications of androgen deprivation therapy in the older man.

authors

- Mohile SG
- Mustian K
- Bylow K
- Hall W
- Dale W

journal: Critical reviews in oncology/hematology

journalAbbr: Crit Rev Oncol Hematol

year: 2009

pubdate: 2009 Jun

volume: 70

issue: 3

pages: 235-55

abstract: Prostate cancer is the most common malignancy in older men. With the aging of the population, the number of older men with prostate cancer will grow rapidly. Androgen deprivation therapy (ADT) is the mainstay of treatment for men with systemic disease and is increasingly utilized as primary therapy or in combination with other therapies for localized disease. Side effects of therapy are multifold and include hot flashes, osteoporosis, and adverse psychological and metabolic effects. Recent research has illustrated that ADT can negatively impact the functional, cognitive, and physical performance of older men. Patients with prostate cancer, despite recurrence of the disease, have a long life expectancy and may be subjected to the side effects of ADT for many years. This review highlights the complications of ADT and approaches to management. We also provide recommendations for assessment and management of ADT complications among the most vulnerable and frail older male patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3074615/

Record 1511

pmcid: PMC2705310

pmid: 20190910

doi: 10.1177/229255030901700105

title: Metastatic cutaneous breast carcinoma: A case report and review of the literature.

authors

- Nava G
- Greer K
- Patterson J
- Lin KY

journal: The Canadian journal of plastic surgery = Journal canadien de chirurgie plastique

journalAbbr: Can J Plast Surg

year: 2009

pubdate: 2009 Spring

volume: 17

issue: 1

pages: 25-7

abstract: Cutaneous metastatic carcinoma, also known as carcinoma erysipeloides, is an unusual clinical finding. In women, it is most frequently found in association with breast cancer rather than other visceral malignancies. Breast cancer patients can present with cutaneous manifestations of breast disease at the time of their initial diagnosis; however, cutaneous metastases more often present well after the initial diagnosis and treatment of the breast disease. Prompt recognition of skin metastases by the plastic surgeon, especially during the delayed setting of reconstructive surgery following mastectomy, allows for earlier intervention in treating the systemic spread of the disease. A clinical case of carcinoma erysipeloides presenting in this delayed setting is outlined in the present report, along with a review of the literature.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2705310/

Record 1512

pmcid: PMC2743211

pmid: 19763205

doi:

title: Misdiagnosed Bipolar Disorder Reveals Itself to be Posttraumatic Stress Disorder with Comorbid Pseudotumor Cerebri: A Case Report.

authors

- Salzbrenner S
- Conaway E

journal: Psychiatry (Edgmont (Pa. : Township))

journalAbbr: Psychiatry (Edgmont)

year: 2009

pubdate: 2009 Aug

volume: 6

issue: 8

pages: 29-32

abstract: We present the case of a patient with a reported history of bipolar II and borderline personality disorders who presented to our inpatient psychiatry department following a suicidal gesture. We determined that she was not suffering from bipolar disorder at all, and we diagnosed her with posttraumatic stress disorder and pseudotumor cerebri. This paper describes the overlap of symptoms of bipolar disorder and posttraumatic stress disorder, which may lead to an incorrect diagnosis. Additionally, the patient had the complicating factor of comorbid pseudotumor cerebri, which we feel contributed to her psychiatric symptoms. Once the patient was properly diagnosed and placed on appropriate treatment, she responded well.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2743211/

Record 1513

pmcid: PMC2779961

pmid: 19881132

doi: 10.4103/0970-1591.56204

title: Patient-doctor relationship: Changing perspectives and medical litigation.

authors

- Ganesh K

journal: Indian journal of urology : IJU : journal of the Urological Society of India

journalAbbr: Indian J Urol

year: 2009

pubdate: 2009 Jul

volume: 25

issue: 3

pages: 356-60

abstract: The patient doctor relational dimer has become complex with the hierarchical or fiduciary manner changing to an equal or un equal relationship. Trust and control are interchangeable, leading to increased patient requirements for disclosure and expectations of a cafeteria approach in diagnoses and management of his/her bodily condition. From any mismatch, there is a potential for medical litigation. In this context, the rise of global consumerism, the explosion of information available on the internet, and the changed manner of the medical profession from being shrouded in mystic / ceremony to trifurcation of medical services to doctoral diagnoses and management, ancillary pharmacy industry, and paramedical services like nursing, counselling and the new age quackery have contributed to this dimer.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2779961/

Record 1514

pmcid: PMC2911830

pmid: 20740101

doi: 10.7812/TPP/08-079

title: Peripartum cardiomyopathy: case reports.

authors

- Wang M

journal: The Permanente journal

journalAbbr: Perm J

year: 2009

pubdate: 2009 Fall

volume: 13

issue: 4

pages: 42-5

abstract: Peripartum cardiomyopathy (PPCM) is a dilated cardiomyopathy defined as systolic cardiac heart failure in the last month of pregnancy or within five months of delivery. PPCM, which affects thousands of women each year in the US, was first described in the 1800s, yet its etiology is still unclear. Its diagnosis is often delayed because its symptoms closely resemble those within the normal spectrum of pregnancy and the postpartum period. When PPCM is misdiagnosed or its diagnosis is delayed, the consequences for patients are deadly: The disorder carries a high mortality rate.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2911830/

Record 1515

pmcid: PMC2778792

pmid: 19956638

doi: 10.1155/2009/578610

title: Persistent hyperprolactinemia and bilateral galactocele in a male infant.

authors

- Tomasi PA
- Fanciulli G
- Casti T
- Delitala G

journal: International journal of pediatric endocrinology

journalAbbr: Int J Pediatr Endocrinol

year: 2009

pubdate: 2009

volume: 2009

issue:

pages: 578610

abstract: Galactocele is a benign breast lesion, usually occurring in nursing women. This lesion is a rare cause

of breast enlargement in children. In this paper we describe the case of an infant with hyperprolactinemia (which persisted throughout 15 years of clinical observation) and bilateral galactocele. We speculate that a congenital midline defect in our patient might have impaired the normal dopaminergic inhibitory tone on pituitary lactotroph cells, thus leading to an increased prolactin secretion by the pituitary gland; this, in turn, might have favored the development of the galactocele.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2778792/

Record 1516

pmcid: PMC2797738

pmid: 19881102

doi: 10.4103/0971-3026.57207

title: Pictorial essay: Mammography of the male breast.

authors

- Popli MB
- Popli V
- Bahl P
- Solanki Y

journal: The Indian journal of radiology & imaging

journalAbbr: Indian J Radiol Imaging

year: 2009

pubdate: 2009 Oct-Dec

volume: 19

issue: 4

pages: 278-81

abstract: Mammography is an imaging modality that is widely perceived to be of use only in women for the detection and diagnosis of breast pathologies. Here, we present a pictorial essay on the mammographic spectrum of male breast pathologies.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2797738/

Record 1517

pmcid: PMC3029298

pmid: 21931579

doi: 10.1136/bcr.03.2009.1677

title: Precocious puberty and a sellar mass.

authors

- Bhansali A
- Jayaprakash P
- Dutta P
- Walia R
- Ravikumar P

journal: BMJ case reports

journalAbbr: BMJ Case Rep

year: 2009

pubdate: 2009

volume: 2009

issue:

pages:

abstract: Precocious puberty in a child less than 3 years of age is usually gonadotropin dependent and is often associated with extrasellar central nervous system disorders. The present report describes the case of a 2½-year-old girl who presented with gonadotropin-independent precocious puberty and had a sellar mass. She was referred for neurosurgical intervention to our institute. On investigation, she was found to have primary hypothyroidism of autoimmune aetiology, which is rarely associated with precocious puberty and sellar mass due to thyrotroph hyperplasia. She was successfully treated with levothyroxine, and with this secondary sexual characteristics progressively regressed and the sellar mass disappeared.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3029298/

Record 1518

pmcid: PMC2697509

pmid: 19436604

doi:

title: Progesterone for the prevention of preterm birth: indications, when to initiate, efficacy and safety.

authors

- How HY
- Sibai BM

journal: Therapeutics and clinical risk management

journalAbbr: Ther Clin Risk Manag

year: 2009

pubdate: 2009 Feb

volume: 5

issue: 1

pages: 55-64

abstract: Preterm birth is the leading cause of neonatal mortality and morbidity and long-term disability of non-anomalous infants. Previous studies have identified a prior early spontaneous preterm birth as the risk factor with the highest predictive value for recurrence. Two recent double blind randomized placebo controlled trials reported lower preterm birth rate with the use of either intramuscular 17 alpha-hydroxyprogesterone caproate (IM 17OHP-C) or intravaginal micronized progesterone suppositories in women at risk for preterm delivery. However, it is still unclear which high-risk women would truly benefit from this treatment in a general clinical setting and whether socio-cultural, racial and genetic differences play a role in patient's response to supplemental progesterone. In addition the patient's acceptance of such recommendation is also in question. More research is still required on identification of at risk group, the optimal gestational age at initiation, mode of administration, dose of progesterone and long-term safety.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2697509/

Record 1519

pmcid: PMC3600411

pmid: 19745648

doi: 10.1097/JCP.0b013e3181b2abf2

title: Randomized, double-blind, placebo-controlled trial of Cimicifuga racemosa (black cohosh) in women with anxiety disorder due to menopause.

authors

- Amsterdam JD
- Yao Y
- Mao JJ
- Soeller I
- Rockwell K
- Shults J

journal: Journal of clinical psychopharmacology

journalAbbr: J Clin Psychopharmacol

year: 2009

pubdate: 2009 Oct

volume: 29

issue: 5

pages: 478-83

abstract: OBJECTIVE: We conducted a randomized, double-blind, placebo-controlled, parallel group trial of the efficacy and tolerability of Cimicifuga racemosa (black cohosh) extract for the treatment of anxiety disorder due to menopause. We hypothesized that black cohosh would be superior to placebo in reducing anxiety symptoms of menopause, with a comparable tolerability profile to placebo. MATERIALS AND METHODS: Subjects were randomized to therapy with either pharmaceutical-grade black cohosh extract (n = 15) or placebo (n = 13) for up to 12 weeks. The primary outcome measure was changed over time in total Hamilton Anxiety Rating Scale (HAM-A) scores. Secondary outcomes included a change in scores on the Beck Anxiety Inventory, Green Climacteric Scale (GCS), and Psychological General Well-Being Index (PGWBI) and the proportion of patients with a change of 50% or higher in baseline HAM-A scores. RESULTS: There was neither a significant group difference in change over time in total HAM-A scores (P = 0.294) nor a group difference in the proportion of subjects with a reduction of 50% or higher in baseline HAM-A scores at study end point (P = 0.79). There was a significantly greater reduction in the total GCS scores during placebo (vs black cohosh; P = 0.035) but no group difference in change over time in the GCS subscale scores or in the PGWBI (P = 0.140). One subject (3.6%) taking black cohosh discontinued treatment because of adverse events. CONCLUSIONS: We found no statistically significant anxiolytic effect of black cohosh (vs placebo). However, small sample size, choice of black cohosh preparation, and dosage used may have been limiting factors producing negative results.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3600411/

Record 1520

pmcid: PMC2798933

pmid: 20309431

doi: 10.1007/s12308-009-0043-y

title: Rare lymphoid malignancies of the breast: a report of two cases illustrating potential diagnostic pitfalls.

authors

- Farkash EA
- Ferry JA
- Harris NL
- Hochberg EP
- Takvorian RW

- Zuckerman DS
- Sohani AR

journal: Journal of hematopathology

journalAbbr: J Hematop

year: 2009

pubdate: 2009 Aug 20

volume: 2

issue: 4

pages: 237-44

abstract: Breast involvement by lymphoma is uncommon and poses challenges in diagnosis. Lymphomas may clinically, radiologically, and morphologically mimic both benign and neoplastic conditions. We describe two cases of lymphoid malignancies predominantly involving the breast, both presenting diagnostic dilemmas. The first case, ALK-negative anaplastic large-cell lymphoma involving a seroma associated with a breast implant, is an emerging clinicopathologic entity. Anaplastic large-cell lymphoma has been identified in association with breast implants and seroma formation relatively recently. The second case, hairy cell leukemia involving the breast and ipsilateral axillary sentinel lymph node, is, to our knowledge, the first reported case of hairy cell leukemia involving the breast at the time of diagnosis. While a localized bone lesion was present at time of diagnosis, bone marrow involvement was relatively mild in comparison to that seen in the breast and lymph node. In the first case, lymphoma occurred in a clinical setting where malignancy was unsuspected, highlighting the importance of careful morphologic evaluation of paucicellular samples, as well as awareness of rare clinicopathologic entities, in avoiding a misdiagnosis of a benign inflammatory infiltrate. In the second case, the lymphoid neoplasm exhibited classic morphologic and immunophenotypic features, but presented at an unusual site of involvement. Knowledge of the patient's concurrent diagnosis of hairy cell leukemia involving the bone marrow and bone helped avoid a misdiagnosis of carcinoma rather than lymphoma.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2798933/

Record 1521

pmcid: PMC2902995

pmid: 18688706

doi: 10.1007/s10461-008-9444-7

title: Self-perception of body fat changes and HAART adherence in the Women's Interagency HIV Study.

authors

- Plankey M
- Bacchetti P
- Jin C
- Grimes B
- Hyman C
- Cohen M
- Howard AA
- Tien PC

journal: AIDS and behavior

journalAbbr: AIDS Behav

year: 2009

pubdate: 2009 Feb

volume: 13

issue: 1

pages: 53-9

abstract: To determine the association of self-perceived fat gain or fat loss in central and peripheral body sites with adherence to highly active antiretroviral therapy (HAART) in HIV-seropositive women. 1,671 women from the Women's Interagency HIV Study who reported HAART use between April 1999 and March 2006 were studied. Adherence was defined as report of taking HAART >/= 95% of the time during the prior 6 months. Participant report of any increase or decrease in the chest, abdomen, or upper back in the prior 6 months defined central fat gain and central fat loss, respectively. Report of any increase or decrease in the face, arms, legs or buttocks in the prior 6 months defined peripheral fat gain or peripheral fat loss. Younger age, being African-American (vs. White non-Hispanic), a history of IDU, higher HIV RNA at the previous visit, and alcohol consumption were significant predictors of HAART non-adherence (P < 0.05). After multivariate adjustment, self-perception of central fat gain was associated with a 1.5-fold increased odds of HAART non-adherence compared to no change. Self-perception of fat gain in the abdomen was the strongest predictor of HAART non-adherence when the individual body sites were studied. Women who perceive central fat gain particularly in the abdomen are at risk for decreased adherence to HAART despite recent evidence to suggest that HIV and specific antiretroviral drugs are more commonly associated with fat loss than fat gain.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2902995/

Record 1522

pmcid: PMC3033154

pmid: 21415985

doi: 10.4084/MJHID.2009.003

title: Thalassaemia and aberrations of growth and puberty.

authors

- Kyriakou A
- Skordis N

journal: Mediterranean journal of hematology and infectious diseases

journalAbbr: Mediterr J Hematol Infect Dis

year: 2009

pubdate: 2009 Jul 27

volume: 1

issue: 1

pages: e2009003

abstract: Endocrine dysfunction in Thalassaemia major (TM) is a common and disturbing complication, which requires prompt recognition and treatment. The contribution of the underlying molecular defect in TM to the development of endocrinopathies is significant because the patients with the more severe genetic defects have a greater rate of iron loading through higher red cell consumption. TM patients frequently present delay of growth and puberty with reduction of final height. The pathogenesis of growth failure is multifactorial and is mainly due to chronic anemia and hypoxia, chronic liver disease, zinc and folic acid deficiency, iron overload, intensive use of chelating agents, emotional factors, and endocrinopathies (hypogonadism, delayed puberty, hypothyroidism) and GH-IGF-1 axis dysregulation. Although appropriate iron chelation therapy can improve growth and development, TM children and adolescents treated intensively with desferrioxamine remain short as well, showing body disproportion between the upper and lower body segment. Body disproportion is independent of pubertal or prepubertal period of greater height gain. Treatment with recombinant GH (rhGH) is recommended when GH deficiency is established, and even so, the therapeutic response is often non satisfactory. Growth acceleration is mostly promoted with sex steroids in children with associated pubertal delay. Sexual complications in TM, which include Delayed Puberty, Arrested Puberty and Hypogonadism, present the commonest endocrine complication. Iron deposition on gonadotroph cells of the pituitary leads to disruption of

gonadotrophin production which is proven by the poor response of FSH and LH to GnRH stimulation. In the majority of patients gonadal function is normal as most women with Amenorrhea are capable of achieving pregnancy with hormonal treatment and similarly men with azoospermia become fathers. Secondary Hypogonadism appears later in life, and is manifested in women as Secondary Amenorrhea and in men as decline in sexual drive and azzoospermia. The damage to the hypothalamus and pituitary is progressive, even when intensive chelating therapy is given and the appearance of Hypogonadism in both sexes is often unavoidable. Close follow up and proper management is crucial for every patient with TM. Early recognition of growth disturbance and prevention of hypogonadism by early and judicious chelation therapy is mandatory for the improvement of their quality of life. Patients with TM can now live a better life due to modern advances in their medical care and our better understanding in the pathogenesis, manifestation and prevention of endocrine complications.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3033154/

Record 1523

pmcid: PMC2941655

pmid: 20877664

doi: 10.1159/000226284

title: The Diagnostic Accuracy of Mammography and Ultrasound in the Evaluation of Male Breast Disease: A New Algorithm.

authors

- Adibelli ZH
- Oztekin O
- Postaci H
- Uslu A

journal: Breast care (Basel, Switzerland)

journalAbbr: Breast Care (Basel)

year: 2009

pubdate: 2009

volume: 4

issue: 4

pages: 255-259

abstract: SUMMARY: BACKGROUND: The purpose of this study was to define the diagnostic accuracy of mammography and ultrasound in the evaluation of male breast disease, and to suggest a diagnostic protocol for male breast disease. MATERIAL AND METHODS: We retrospectively reviewed clinical, radiographic, and pathologic records of 75 patients. Breast Imaging Reporting and Data System (BI-RADS) category 4-5 mammograms and ultrasonograms were suggested as suspicious for malignancy. RESULTS: Of the 75 patients, 23 (31%) were considered to have suspicious lesions by mammography and/or ultrasonography. 13 of the patients were shown to have breast cancer. The remaining 52 (69%) were referred for biopsy by clinicians; all of the biopsy specimens were benign (gynecomastia). The accuracy data of mammography and ultrasonography are: sensitivity, 69 and 100%; specificity, 87 and 97%; positive predictive value, 53 and 87%; negative predictive value, 93 and 100%; and accuracy, 84 and 97%, respectively. CONCLUSION: We suggest a new diagnostic algorithm for the evaluation of male breast disease in which ultrasonography may be used to evaluate palpable abnormalities as the first diagnostic tool of choice. To use and to trust imaging would decrease the number of false-positive biopsies that would be generated by physical examination alone.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2941655/

Record 1524

pmcid: PMC3098121

pmid: 19373620

doi: 10.1080/10673220902891836

title: Update on research and treatment of premenstrual dysphoric disorder.

authors

- Cunningham J
- Yonkers KA
- O'Brien S
- Eriksson E

journal: Harvard review of psychiatry

journalAbbr: Harv Rev Psychiatry

year: 2009

pubdate: 2009

volume: 17

issue: 2

pages: 120-37

abstract: Many women in their reproductive years experience some mood, behavioral. or physical symptoms in the week prior to menses. Variability exists in the level of symptom burden in that some women experience mild symptoms, whereas a small minority experience severe and debilitating symptoms. For an estimated 5%-8% of premenopausal women, work or social functioning are affected by severe premenstrual syndrome. Many women in this group meet diagnostic criteria for premenstrual dysphoric disorder (PMDD). Among women who suffer from PMDD, mood and behavioral symptoms such as irritability, depressed mood, tension, and labile mood dominate. Somatic complaints, including breast tenderness and bloating, also can prove disruptive to women's overall functioning and quality of life. Recent evidence suggests that individual sensitivity to cyclical variations in levels of gonadal hormones may predispose certain women to experience these mood, behavioral, and somatic symptoms. Treatments include: antidepressants of the serotonin reuptake inhibitor class, taken intermittently or throughout the menstrual cycle; medications that suppress ovarian cyclicity; and newer oral contraceptives with novel progestins.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3098121/

Record 1525

pmcid: PMC2211356

pmid: 18227451

doi: 10.1503/cmaj.071679

title: A new predictor for type 2 diabetes?

authors

- Sorisky A

journal: CMAJ : Canadian Medical Association journal = journal de l'Association medicale canadienne

journalAbbr: CMAJ

year: 2008

pubdate: 2008 Jan 29

volume: 178

issue: 3

pages: 313-5

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2211356/

Record 1526

pmcid: PMC2825138

pmid: 20174542

doi:

title: Breast augmentation.

authors

- Ramachandran K

journal: Indian journal of plastic surgery : official publication of the Association of Plastic Surgeons of India

journalAbbr: Indian J Plast Surg

year: 2008

pubdate: 2008 Oct

volume: 41

issue: Suppl

pages: S41-7

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2825138/

Record 1527

pmcid: PMC2615356

pmid: 18972596

doi: 10.3349/ymj.2008.49.5.757

title: Diagnosis of pseudoangiomatous stromal hyperplasia of the breast: ultrasonography findings and different biopsy methods.

authors

- Choi YJ
- Ko EY
- Kook S

journal: Yonsei medical journal

journalAbbr: Yonsei Med J

year: 2008

pubdate: 2008 Oct 31

volume: 49

issue: 5

pages: 757-64

abstract: PURPOSE: Pseudoangiomatous stromal hyperplasia (PASH) of the breast is a rare, benign condition that can be mistaken as a fibroadenoma on an ultrasound examination or as a low-grade angiosarcoma on a histological examination. The objective of this study was to evaluate the ultrasound features and to present biopsy methods to correctly identify PASH. PATIENTS AND METHODS: We retrospectively reviewed the data of 55 women who were diagnosed with PASH of the breast. Ultrasound features were evaluated according to

the Breast Imaging Reporting and Data System (BI-RADS; American College of Radiology). The diagnostic ability of different biopsy methods such as core needle biopsy, vacuum-assisted biopsy and excisional biopsy were analyzed with the final histopathological results of surgical specimens. RESULTS: PASH presented as a circumscribed solid mass, with hypoechoic texture with or without heterogeneity, and a parallel orientation. The features of small, internal cysts or vascular channels and no calcifications can be used to differentiate the lesions from fibroadenomas. A core needle biopsy misdiagnosed PASH in 13 cases out of 28 cases and vacuum-assisted biopsy correctly identified PASH in all 3 cases. CONCLUSION: Ultrasound features of PASH should be noted when performing a biopsy. For inconclusive cases of PASH, an excisional biopsy followed by an initial core biopsy should be performed.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2615356/

Record 1528

pmcid: PMC2574732

pmid: 18446090

doi: 10.1097/gme.0b013e318167b8f2

title: Effects of a high daily dose of soy isoflavones on DNA damage, apoptosis, and estrogenic outcomes in healthy postmenopausal women: a phase I clinical trial.

authors

- Pop EA
- Fischer LM
- Coan AD
- Gitzinger M
- Nakamura J
- Zeisel SH

journal: Menopause (New York, N.Y.)

journalAbbr: Menopause

year: 2008

pubdate: 2008 Jul-Aug

volume: 15

issue: 4 Pt 1

pages: 684-92

abstract: OBJECTIVE: A phase I double-blind clinical trial was conducted to evaluate the effects of a high oral dose of soy isoflavones administered daily for 84 days to healthy postmenopausal women. Principal outcome measures included DNA damage, apoptosis, and changes indicative of estrogenic stimulation. DESIGN: After eligibility and equol-producer status were determined, stratified randomization was used to assign women to the isoflavone (active) or placebo group. Of the 30 women who completed the study, 18 were in the active group. DNA damage was assessed via COMET and apurinic/apyrimidinic site assays in lymphocytes. Apoptosis was evaluated via terminal deoxynucleotidyl transferase-mediated dUTP nick end labeling and activated caspase-3 assays in lymphocytes. Estrogenic/antiestrogenic effects were assessed using a self-report questionnaire and by assaying for estrogen, follicle-stimulating hormone, luteinizing hormone, and sex hormone-binding globulin in blood. RESULTS: In treated postmenopausal women, there was no indication that high doses of soy isoflavones caused DNA strand breakage, increased apurinic/apyrimidinic sites, or increased apoptosis in peripheral lymphocytes. There were no significant changes in mean values for estrogenic effects or other laboratory measurements. Very few adverse events occurred, and the only drug-related adverse events were mild or grade 1 in severity. CONCLUSIONS: Unconjugated soy isoflavones appear to be safe and well tolerated in healthy postmenopausal women at doses of 900 mg/day.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2574732/

Record 1529

pmcid: PMC2635619

pmid: 19330061

doi: 10.3390/ijms9112062

title: Fumonisins, trichothecenes and zearalenone in cereals.

authors

- Yazar S
- Omurtag GZ

journal: International journal of molecular sciences

journalAbbr: Int J Mol Sci

year: 2008

pubdate: 2008 Nov

volume: 9

issue: 11

pages: 2062-2090

abstract: Fumonisins are phytotoxic mycotoxins which are synthesized by various species of the fungal genus Fusarium such as Fusarium verticillioides (Sacc.) Nirenberg (ex F.moniliforme Sheldon) and Fusarium proliferatum. The trichothecene (TC) mycotoxins are secondary metabolites produce by species that belong to several fungal genera, especially Fusarium, Stachybotrys, Trichothecium, Trichoderma, Memnoniella and Myrothecium. Fusarium mycotoxins are widely dispersed in cereals and their products. Zearalenone (ZEA) is an estrogenic compound produced by Fusarium spp. such as F. graminearum and F. culmorum. Fumonisins, the TCs and ZEA are hazardous for human and animal health. Contamination with TCs causes a number of illnesses in human and animal such as decrease in food consumption (anorexia), depression or inhibition on immune system function and haematoxicity. The purpose of this paper is to give a review of the papers published on the field of fumonisin, TC and ZEA mycotoxins in cereals consumed in the world.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2635619/

Record 1530

pmcid: PMC2276281

pmid: 18369226

doi: 10.1136/bmj.39511.493391.BE

title: Gynaecomastia and breast cancer in men.

authors

- Niewoehner CB
- Schorer AE

journal: BMJ (Clinical research ed.)

journalAbbr: BMJ

year: 2008

pubdate: 2008 Mar 29

volume: 336

issue: 7646

pages: 709-13

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2276281/

Record 1531

pmcid: PMC2776678

pmid: 18586190

doi: 10.1016/j.jmwh.2008.01.006

title: Hormonal contraception and HIV-positive women: metabolic concerns and management strategies.

authors

- Womack J
- Richman S
- Tien PC
- Grey M
- Williams A

journal: Journal of midwifery & women's health

journalAbbr: J Midwifery Womens Health

year: 2008

pubdate: 2008 Jul-Aug

volume: 53

issue: 4

pages: 362-75

abstract: As HIV-positive women live longer lives, and as testing for HIV becomes more routine, clinicians can expect to see more HIV-positive women in their practices. The need to be aware of management issues particular to this population becomes increasingly important. Metabolic dysregulation is a common, long-term complication associated with HIV and is one of the most difficult to manage. Hormonal contraception also is associated with metabolic dysregulation. As more HIV-positive women choose long-term, reversible contraception, the potential for concomitant and additive side effects, and the need for careful, proactive management strategies to avoid these complications, will become more important. This article reviews research detailing the metabolic dysfunction associated with hormonal contraception and with HIV-seropositivity. It highlights reasons for concern regarding the potential, although as yet theoretical, increased risk for metabolic dysfunction when hormonal contraception is used in the presence of HIV. Suggestions for management strategies for women living with HIV who choose to use hormonal contraception are presented. These strategies should be viewed as suggestions for management until substantitive research becomes available.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2776678/

Record 1532

pmcid: PMC4897324

pmid: 27303554

doi: 10.2484/rcr.v3i4.214

title: Intracystic Papillary Carcinoma in a Man with Gynecomastia.

authors

- Johnson JB
- Emory TH

journal: Radiology case reports

journalAbbr: Radiol Case Rep

year: 2008

pubdate: 2008

volume: 3

issue: 4

pages: 214

abstract: Male breast cancer is a rare disease in the male breast whereas gynecomastia is quite common. An elevation of the estrogen-to-androgen ratio increases the risk of both of these diseases. However, a connection between gynecomastia and subsequent breast cancer development is controversial and unclear. Imaging studies including mammography and ultrasound provide valuable information in leading to a correct diagnosis. Traditionally, intracystic papillary carcinoma, also known as encapsulated papillary carcinoma, has been considered a form of ductal carcinoma in situ. Recent immunohistochemical studies, demonstrating an absence of myothelium, in many cases would be more compatible with the diagnosis of invasive malignancy. However, intracystic papillary carcinoma holds a favorable prognosis with local therapy alone. We report a case of intracystic papillary carcinoma in a male patient with long-standing gynecomastia diagnosed eight years prior by mammography. The patient presented with a breast lump on both occasions. Current work-up consisted of both mammography and ultrasound. Ultrasound provided key information revealing a complex mass requiring further evaluation. Ultrasound-guided core needle biopsy revealed intracystic papillary carcinoma with confirmation upon surgical excision.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4897324/

Record 1533

pmcid: PMC2656416

pmid: 18820648

doi: 10.1038/oby.2008.418

title: Long-term effects of large-volume liposuction on metabolic risk factors for coronary heart disease.

authors

- Mohammed BS
- Cohen S
- Reeds D
- Young VL
- Klein S

journal: Obesity (Silver Spring, Md.)

journalAbbr: Obesity (Silver Spring)

year: 2008

pubdate: 2008 Dec

volume: 16

issue: 12

pages: 2648-51

abstract: Abdominal obesity is associated with metabolic risk factors for coronary heart disease (CHD). Although we previously found that using liposuction surgery to remove abdominal subcutaneous adipose tissue (SAT) did not result in metabolic benefits, it is possible that postoperative inflammation masked the beneficial effects. Therefore, this study provides a long-term evaluation of a cohort of subjects from our original study. Body composition and metabolic risk factors for CHD, including oral glucose tolerance, insulin resistance, plasma lipid profile, and blood pressure were evaluated in seven obese (39 +/- 2 kg/m(2)) women before and at 10, 27, and

84-208 weeks after large-volume liposuction. Liposuction surgery removed 9.4 +/- 1.8 kg of body fat (16 +/- 2% of total fat mass; 6.1 +/- 1.4 kg decrease in body weight), primarily from abdominal SAT; body composition and weight remained the same from 10 through 84-208 weeks. Metabolic endpoints (oral glucose tolerance, homeostasis model assessment of insulin resistance, blood pressure and plasma triglyceride (TG), high-density lipoprotein (HDL)-cholesterol, and low-density lipoprotein (LDL)-cholesterol concentrations) obtained at 10 through 208 weeks were not different from baseline and did not change over time. These data demonstrate that removal of a large amount of abdominal SAT by using liposuction does not improve CHD metabolic risk factors associated with abdominal obesity, despite a long-term reduction in body fat.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2656416/

Record 1534

pmcid: PMC2621403

pmid: 19209283

doi: 10.2147/tcrm.s3285

title: Lopinavir/ritonavir in the treatment of HIV-1 infection: a review.

authors

- Chandwani A
- Shuter J

journal: Therapeutics and clinical risk management

journalAbbr: Ther Clin Risk Manag

year: 2008

pubdate: 2008 Oct

volume: 4

issue: 5

pages: 1023-33

abstract: Lopinavir/ritonavir is the first and only coformulated HIV-1 protease inhibitor (PI). Large clinical trials have demonstrated lopinavir/ritonavir's clinical efficacy in both antiretroviral-naïve and -experienced patients. The immunologic and virologic benefits of treatment with this agent have been proven in HIV-infected adults, adolescents, and children. Smaller studies support the use of lopinavir/ritonavir monotherapy as a therapeutic option in certain patients. The drug is characterized by a high genetic barrier to resistance, and appears to be more forgiving of non-adherence than earlier, unboosted PIs. The most frequent side effects observed are diarrhea, nausea, and vomiting. These gastrointestinal adverse effects are generally mild to moderate. Metabolic derangements, including hyperlipidemia and glucose intolerance, have also been observed in lopinavir/ritonavir recipients. As the menu of available antiretroviral agents continues to expand, lopinavir/ritonavir remains a proven and effective drug for the treatment of HIV infection.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2621403/

Record 1535

pmcid: PMC2413049

pmid: 18453813

doi: 10.1111/j.1559-4572.2008.07595.x

title: Magnetic resonance imaging for quantifying regional adipose tissue in human immunodeficiency virus-infected persons with the cardiometabolic syndrome.

authors

- Bashir A

- Laciny E
- Lassa-Claxton S
- Yarasheski KE

journal: Journal of the cardiometabolic syndrome

journalAbbr: J Cardiometab Syndr

year: 2008

pubdate: 2008 Spring

volume: 3

issue: 2

pages: 115-8

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2413049/

Record 1536

pmcid: PMC2440380

pmid: 18558006

doi: 10.1186/1477-7819-6-58

title: Male breast cancer: is the scenario changing.

authors

- Contractor KB
- Kaur K
- Rodrigues GS
- Kulkarni DM
- Singhal H

journal: World journal of surgical oncology

journalAbbr: World J Surg Oncol

year: 2008

pubdate: 2008 Jun 16

volume: 6

issue:

pages: 58

abstract: BACKGROUND: The overall incidence of male breast cancer is around 1% of all breast cancers and is on the rise. In this review we aim to present various aspects of male breast cancer with particular emphasis on incidence, risk factors, patho-physiology, treatment, prognostic factors, and outcome. METHODS: Information on all aspects of male breast cancer was gathered from available relevant literature on male breast cancer from the MEDLINE database over the past 32 years from 1975 to 2007. Various reported studies were scrutinized for emerging evidence. Incidence data were also obtained from the IARC, Cancer Mondial database. CONCLUSION: There is a scenario of rising incidence, particularly in urban US, Canada and UK. Even though more data on risk factors is emerging about this disease, more multi-institutional efforts to pool data with large randomized trials to show treatment and survival benefits are needed to support the existing vast emerging knowledge about the disease.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2440380/

Record 1537

pmcid: PMC2430323

pmid: 19119333

doi: 10.1624/105812408X266287

title: Normalizing the changes experienced during each trimester of pregnancy.

authors

- Bonillas CA
- Feehan R

journal: The Journal of perinatal education

journalAbbr: J Perinat Educ

year: 2008

pubdate: 2008 Winter

volume: 17

issue: 1

pages: 39-43

abstract: In this column, the various physical changes that may occur during each trimester of pregnancy are introduced in an activity that involves participants in childbirth education classes or in a waiting room of a prenatal care clinic. This lesson is intended to educate and reassure women of the normalcy of the changes they may experience throughout a pregnancy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2430323/

Record 1538

pmcid: PMC3012441

pmid: 21221191

doi:

title: Paliperidone: the evidence of its therapeutic value in schizophrenia.

authors

- Kantrowitz J
- Citrome L

journal: Core evidence

journalAbbr: Core Evid

year: 2008

pubdate: 2008 Jul 31

volume: 2

issue: 4

pages: 261-71

abstract: INTRODUCTION: Paliperidone, the 9-hydroxy metabolite of risperidone, is a second-generation antipsychotic that was recently approved for the treatment of schizophrenia. It is marketed as an improvement over risperidone, but is likely to be considerably more costly when risperidone is no longer protected by patent. AIMS: To review the evidence for the clinical impact of paliperidone in the treatment of patients with schizophrenia, particularly in contrast to risperidone. EVIDENCE REVIEW: Paliperidone is primarily metabolized and excreted renally, and thus may be of particular utility for patients with hepatic impairment. There is clear

evidence that paliperidone is more efficacious than placebo in reducing the positive and negative symptoms of schizophrenia. In patients with schizophrenia, paliperidone has been shown to stabilize acute psychotic symptoms. There is some evidence that it can prevent relapse in stabilized patients. Studies on the cost effectiveness of paliperidone are needed. Most importantly, there are no trials comparing paliperidone directly with other second-generation antipsychotics. PLACE IN THERAPY: Until direct efficacy and cost effectiveness comparisons are made with risperidone, it is difficult to justify paliperidone use over risperidone. It will become even harder to justify when risperidone becomes available as a less expensive generic medication.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3012441/

Record 1539

pmcid: PMC2730082

pmid: 19750156

doi:

title: Pharmaceutical approval update.

authors

- Goldenberg MM

journal: P & T : a peer-reviewed journal for formulary management

journalAbbr: P T

year: 2008

pubdate: 2008 Mar

volume: 33

issue: 3

pages: 150-81

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2730082/

Record 1540

pmcid: PMC3118460

pmid: 18395582

doi: 10.1016/S0140-6736(08)60527-9

title: Premenstrual syndrome.

authors

- Yonkers KA
- O'Brien PM
- Eriksson E

journal: Lancet (London, England)

journalAbbr: Lancet

year: 2008

pubdate: 2008 Apr 5

volume: 371

issue: 9619

pages: 1200-10

abstract: Most women of reproductive age have some physical discomfort or dysphoria in the weeks before menstruation. Symptoms are often mild, but can be severe enough to substantially affect daily activities. About 5-8% of women thus suffer from severe premenstrual syndrome (PMS); most of these women also meet criteria for premenstrual dysphoric disorder (PMDD). Mood and behavioural symptoms, including irritability, tension, depressed mood, tearfulness, and mood swings, are the most distressing, but somatic complaints, such as breast tenderness and bloating, can also be problematic. We outline theories for the underlying causes of severe PMS, and describe two main methods of treating it: one targeting the hypothalamus-pituitary-ovary axis, and the other targeting brain serotonergic synapses. Fluctuations in gonadal hormone levels trigger the symptoms, and thus interventions that abolish ovarian cyclicity, including long-acting analogues of gonadotropin-releasing hormone (GnRH) or oestradiol (administered as patches or implants), effectively reduce the symptoms, as can some oral contraceptives. The effectiveness of serotonin reuptake inhibitors, taken throughout the cycle or during luteal phases only, is also well established.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3118460/

Record 1541

pmcid: PMC2738334

pmid: 19742215

doi: 10.4103/0019-5545.42398

title: Psychosocial and cultural aspects of pseudocyesis.

authors

- Ibekwe PC
- Achor JU

journal: Indian journal of psychiatry

journalAbbr: Indian J Psychiatry

year: 2008

pubdate: 2008 Apr

volume: 50

issue: 2

pages: 112-6

abstract: Though considered rare in the developed countries, pseudocyesis is fairly common in gynecological practices in Africa. Using a case report and an overview of the literature, this paper posits that the elucidation of the psychosocial and cultural contexts within which a given patient lives can provide a basis for the empathic understanding of the reasons for the development of pseudocyesis. The case underscores the contributions of extreme poverty, relationship instability, and recurrent partner abuse in the enactment of pseudocyesis within a culture that treasures children for economic survival and generational continuity. The awareness of this cultural dimension is considered relevant to effective clinical care.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2738334/

Record 1542

pmcid: PMC3049948

pmid: 18398147

doi: 10.1200/JCO.2007.13.5517

title: Toremifene improves lipid profiles in men receiving androgen-deprivation therapy for prostate cancer: interim analysis of a multicenter phase III study.

authors

- Smith MR

- Malkowicz SB
- Chu F
- Forrest J
- Sieber P
- Barnette KG
- Rodriquez D
- Steiner MS

journal: Journal of clinical oncology : official journal of the American Society of Clinical Oncology

journalAbbr: J Clin Oncol

year: 2008

pubdate: 2008 Apr 10

volume: 26

issue: 11

pages: 1824-9

abstract: PURPOSE: Androgen-deprivation therapy (ADT) is associated with greater risk of incident coronary heart disease and hospital admission for myocardial infarction; treatment-related increases in serum lipids may contribute to greater cardiovascular disease risk. We evaluated the effects of toremifene, a selective estrogen-receptor modulator, on fasting serum lipid levels in men receiving ADT for prostate cancer. PATIENTS AND METHODS: In an ongoing, multicenter, double-blind, placebo-controlled phase III fracture-prevention study, 1,389 men receiving ADT for prostate cancer were randomly assigned to receive toremifene (80 mg/d) or placebo. In this interim analysis of 188 patients, changes in fasting serum lipids from baseline to month 12 were compared between the placebo and toremifene groups. RESULTS: Changes in serum lipids differed significantly between the groups. Mean (+/- SE) total cholesterol decreased by 1.0% +/- 1.7% from baseline to month 12 in the placebo group and decreased by 8.1% +/- 1.4% in the toremifene group (P = .001 for between group comparison). Low-density lipoprotein (LDL) cholesterol increased by 0.8% +/- 2.5% in the placebo group and decreased by 8.2% +/- 2.5% in the toremifene group (P = .003). In contrast, high-density lipoprotein (HDL) cholesterol decreased by 4.9% +/- 1.2% in the placebo group and increased by 0.5% +/- 2.2% in the toremifene group (P = .018). Triglycerides increased by 6.9% +/- 4.2% in the placebo group and decreased by 13.2% +/- 3.6% in the toremifene group (P = .003). CONCLUSION: Toremifene significantly decreased total cholesterol, LDL cholesterol, and triglycerides, and increased HDL cholesterol in men receiving ADT for prostate cancer.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3049948/

Record 1543

pmcid: PMC3090686

pmid: 18001802

doi: 10.1016/j.juro.2007.08.137

title: Toremifene increases bone mineral density in men receiving androgen deprivation therapy for prostate cancer: interim analysis of a multicenter phase 3 clinical study.

authors

- Smith MR
- Malkowicz SB
- Chu F
- Forrest J
- Price D
- Sieber P

- Barnette KG
- Rodriguez D
- Steiner MS

journal: The Journal of urology

journalAbbr: J Urol

year: 2008

pubdate: 2008 Jan

volume: 179

issue: 1

pages: 152-5

abstract: PURPOSE: We evaluated the effects of toremifene on bone mineral density, a surrogate for fracture risk, in men receiving androgen deprivation therapy for prostate cancer. MATERIALS AND METHODS: In an ongoing, multicenter, phase 3 fracture prevention study 1,392 men 50 years or older with prostate cancer receiving androgen deprivation therapy were randomized to 80 mg toremifene per day or placebo. Bone mineral density of the lumbar spine, total hip and femoral neck was assessed using dual energy x-ray absorptiometry. In this planned interim analysis of the first 197 subjects we compared bone mineral density changes from baseline to month 12 between the placebo and toremifene groups. RESULTS: Compared with the placebo group men in the toremifene group had significant increases in bone mineral density at each evaluated skeletal site. Lumbar spine bone mineral density decreased 0.7% in the placebo group and increased 1.6% in the toremifene group (between group comparison p <0.001). Total hip bone mineral density decreased 1.3% in the placebo group and increased 0.7% in the toremifene group (p = 0.001). Femoral neck bone mineral density decreased 1.3% in the placebo group and increased 0.2% in the toremifene group (p = 0.009). Between group differences in the change in bone mineral density from baseline to month 12 were 2.3%, 2.0% and 1.5% for the lumbar spine, total hip and femoral neck, respectively. CONCLUSIONS: Toremifene significantly increased hip and spine bone mineral density in men receiving androgen deprivation therapy for prostate cancer. The effect of toremifene on the fracture risk is being assessed in the ongoing randomized, controlled trial.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3090686/

Record 1544

pmcid: PMC2884857

pmid: 20567689

doi: 10.1055/s-2007-1019143

title: Transumbilical breast augmentation is safe and effective.

authors

- Dowden RV

journal: Seminars in plastic surgery

journalAbbr: Semin Plast Surg

year: 2008

pubdate: 2008 Feb

volume: 22

issue: 1

pages: 51-9

abstract: Transumbilical breast augmentation (TUBA) has proved to be safe and effective. This article highlights the advantages, disadvantages, gives an overview of the technique, discusses complications, and provides factual information to counter false negative information about the procedure.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2884857/

Record 1545

pmcid: PMC5477916

pmid: 28657010

doi: 10.1093/ndtplus/sfn011

title: Unilateral pleural effusion associated with ipsilateral arm and breast oedema: a rare complication of brachiocephalic and SVC stenosis in association with an arteriovenous fistula.

authors

- Campbell G
- Nicholson T
- Mooney A

journal: NDT plus

journalAbbr: NDT Plus

year: 2008

pubdate: 2008 Apr

volume: 1

issue: 2

pages: 131-132

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5477916/

Record 1546

pmcid: PMC2244934

pmid: 18057139

doi: 10.1210/er.2007-0017

title: What can we learn from rodents about prolactin in humans?

authors

- Ben-Jonathan N
- LaPensee CR
- LaPensee EW

journal: Endocrine reviews

journalAbbr: Endocr Rev

year: 2008

pubdate: 2008 Feb

volume: 29

issue: 1

pages: 1-41

abstract: Prolactin (PRL) is a 23-kDa protein hormone that binds to a single-span membrane receptor, a member of the cytokine receptor superfamily, and exerts its action via several interacting signaling pathways. PRL is a multifunctional hormone that affects multiple reproductive and metabolic functions and is also involved

in tumorigenicity. In addition to being a classical pituitary hormone, PRL in humans is produced by many tissues throughout the body where it acts as a cytokine. The objective of this review is to compare and contrast multiple aspects of PRL, from structure to regulation, and from physiology to pathology in rats, mice, and humans. At each juncture, questions are raised whether, or to what extent, data from rodents are relevant to PRL homeostasis in humans. Most current knowledge on PRL has been obtained from studies with rats and, more recently, from the use of transgenic mice. Although this information is indispensable for understanding PRL in human health and disease, there is sufficient disparity in the control of the production, distribution, and physiological functions of PRL among these species to warrant careful and judicial extrapolation to humans.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2244934/

Record 1547

pmcid: PMC2927128

pmid: 20740062

doi: 10.1016/j.dr.2006.11.001

title: Detrimental Psychological Outcomes Associated with Early Pubertal Timing in Adolescent Girls.

authors

- Mendle J
- Turkheimer E
- Emery RE

journal: Developmental review : DR

journalAbbr: Dev Rev

year: 2007

pubdate: 2007 Jun

volume: 27

issue: 2

pages: 151-171

abstract: Though often discussed as though it were a discrete event, puberty comprises one segment of a larger developmental continuum and is notable for rapid transformation across a multitude of domains. Research suggests that an earlier rate of pubertal maturation in girls correlates with a number of detrimental outcomes compared with on-time or later maturation. The present review synthesizes the research on negative psychological sequelae of early pubertal timing in adolescent girls. Emphasis is on three theoretical perspectives by which precocious development is believed to affect the emergence of adverse outcomes: biological, psychosocial, and selection effects.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2927128/

Record 1548

pmcid: PMC7151804

pmid:

doi: 10.1016/B978-032303506-4.10358-X

title: Differential Diagnosis.

authors

journal: Pediatric Clinical Advisor

journalAbbr: Pediatric Clinical Advisor

year: 2007

pubdate: 2007

volume:

issue:

pages: 613-32

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7151804/

Record 1549

pmcid: PMC1934908

pmid: 17587461

doi: 10.1186/1477-7819-5-71

title: Dysgerminoma in three patients with Swyer syndrome.

authors

- Behtash N
- Karimi Zarchi M

journal: World journal of surgical oncology

journalAbbr: World J Surg Oncol

year: 2007

pubdate: 2007 Jun 23

volume: 5

issue:

pages: 71

abstract: BACKGROUND: Dysgerminoma is the most common malignant germ cell tumor of the ovary. This malignancy can be associated with pure gonadal dysgenesis or Swyer syndrome, mixed gonadal dysgenesis and partial gonadal dysgenesis. CASE PRESENTATION: Dysgerminoma developed in 3 phenotypic female patients with 46 XY pure gonadal dysgenesis. All patients presented first with abdominopelvic mass. Laparatomy was done. 46 XY karyotype was made by lymphocyte culture. Then these patients underwent gonadectomy that histopathology results were streak ovaries without evidence for malignancy. Two patients received postoperative adjuvant therapy. CONCLUSION: In Patients with Swyer syndrome the risk of dysgerminoma is high and gonadectomy is recommended. Also 5% of dysgerminomas are discovered in phenotypic female and 46 XY karyotype, thus in adolescent with dysgerminoimas and amenorrhea, karyotype should be done.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1934908/

Record 1550

pmcid: PMC2213888

pmid: 18231613

doi:

title: Estrogenic side effects of androgen deprivation therapy.

authors

- Guise TA
- Oefelein MG
- Eastham JA

- Cookson MS
- Higano CS
- Smith MR

journal: Reviews in urology

journalAbbr: Rev Urol

year: 2007

pubdate: 2007 Fall

volume: 9

issue: 4

pages: 163-80

abstract: Androgen deprivation therapy (ADT) is part of standard therapy for locally advanced or metastatic prostate cancer and is frequently used in men with a rising prostate-specific antigen following radical prostatectomy or radiation therapy. In some men, ADT may be administered for years or even decades. The intended therapeutic effect of ADT is testosterone deficiency. Because estrogen is a normal metabolite of testosterone, ADT also results in estrogen deficiency. ADT has a variety of adverse effects, many of which are primarily related to estrogen deficiency. Bone mineral density may decrease by 4% to 13% per year in men receiving ADT. The fracture rate for patients on ADT averages 5% to 8% per year of therapy. Hot flashes, gynecomastia, and breast tenderness are common side effects associated with ADT. In the clinic, minimum baseline testing should include weight measurement, blood pressure reading, and fasting lipid panel and serum glucose tests. Currently, there are no large outcome trials in men on ADT testing the available therapies for adverse effects. No therapies are specifically approved for treatment of adverse effects in men on ADT. Although some therapies can be used for a single indication (based upon small studies), there is currently no agent to treat the multiple estrogenic side effects of ADT.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2213888/

Record 1551

pmcid: PMC2685268

pmid: 18044182

doi:

title: Eszopiclone for late-life insomnia.

authors

- McCrae CS
- Ross A
- Stripling A
- Dautovich ND

journal: Clinical interventions in aging

journalAbbr: Clin Interv Aging

year: 2007

pubdate: 2007

volume: 2

issue: 3

pages: 313-26

abstract: Insomnia, the most common sleep disturbance in later life, affects 20%-50% of older adults. Eszopiclone, a short-acting nonbenzodiazepine hypnotic agent developed for the treatment of insomnia, has

been available in Europe since 1992 and in the US since 2005. Although not yet evaluated for transient insomnia in older adults, eszopiclone has been shown to be safe and efficacious for short-term treatment (2 weeks) of chronic, primary insomnia in older adults (64-91 years). Clinical studies in younger adults (mean = 44 years) have shown eszopiclone can be used for 6-12 months without evidence of problems. Because the oldest participant in these longer-term trials was 69, it not known whether eszopiclone is effective for older adults [particularly the old old (75-84 years) and oldest old (85+)] when used over longer periods. This is unfortunate, because older individuals frequently suffer from chronic insomnia. Cognitive-behavioral therapy for insomnia, which effectively targets the behavioral factors that maintain chronic insomnia, represents an attractive treatment alternative or adjuvant to eszopiclone for older adults. To date, no studies have compared eszopiclone to other hypnotic medications or to nonpharmacological interventions, such as cognitive-behavioral therapy for insomnia, in older adults. All of the clinical trials reported herein were funded by Sepracor. This paper provides an overview of the literature on eszopiclone with special emphasis on its use for the treatment of late-life insomnia. Specific topics covered include pharmacology, pharmacodynamics, pharmacokinetics, clinical trial data, adverse events, drug interactions, tolerance/dependence, and economics/cost considerations for older adults.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2685268/

Record 1552

pmcid: PMC2083432

pmid: 17337691

doi: 10.1136/adc.2006.110528

title: Is the topical application of oestrogen cream an effective intervention in girls suffering from labial adhesions?

authors

- Tebruegge M
- Misra I
- Nerminathan V

journal: Archives of disease in childhood

journalAbbr: Arch Dis Child

year: 2007

pubdate: 2007 Mar

volume: 92

issue: 3

pages: 268-71

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2083432/

Record 1553

pmcid: PMC2686040

pmid: 19554126

doi: 10.1177/229255030701500107

title: Late unilateral breast enlargement after insertion of silicone gel implants: A histopathological study.

authors

- Peters W
- Fornasier V

journal: The Canadian journal of plastic surgery = Journal canadien de chirurgie plastique

journalAbbr: Can J Plast Surg

year: 2007

pubdate: 2007 Spring

volume: 15

issue: 1

pages: 19-28

abstract: Late unilateral breast enlargement after the insertion of silicone gel breast implants is a very rare phenomenon. The present study reports five women who presented with this finding over the past 20 years. Three of these patients presented with late unilateral hematomas, which developed nine, 12 and 14 years, respectively, after initial breast augmentation, in the absence of any known trauma. These patients presented for treatment one, four and 12 months, respectively, after their breast enlargements initially appeared. Two of these patients had developed chronic expanding hematomas. Extensive histopathological analyses of the capsules of all three patients provided explanations for the etiologies and progressions of the findings in these patients. In each of the three patients, the etiology of the hematoma was consistent with erosion of a capsular artery. Numerous large vessels were seen within the wall of the capsules. In each case, there was a class IV capsular contracture, which could have increased the friction of the intact implant against the capsule, and there was both old (hemosiderin deposits) and new bleeding into the pocket from the capsules. This supports the concept that numerous episodes of bleeding had occurred in each case. Histopathology also demonstrated the progression of the hematomas. After four weeks, only liquefied hematoma was present, while after four months, there were both liquefied hematoma and blood clotting. The hematoma on the surface of the capsule was becoming organized peripherally, but not centrally. After 12 months, the hematoma was becoming very well organized compared with the hematoma at four months. In one of the two remaining cases, late infection was the cause of the breast enlargement. Histopathology of this capsule showed that the involved capsule was six times as thick as the other side. It also showed edema and infiltration by scattered mononuclear cells, polymorphonuclear cells, and irregular crenated and degenerating nuclei. In the final patient, chronic inflammation appeared to be the cause of the breast enlargement. The histopathology of this capsule was unique. Its inner surface showed re-epithelialization and metaplasia of the ductal epithelium to form stratified squamous epithelium with early surface keratinization. These findings are consistent with synovial metaplasia. Several areas of the fibrous portion of the capsule showed patchy loss of cellular staining with loss of nuclei, indicative of necrosis. This produced a 'washed out' staining appearance. This tissue was paucivascular and fibrotic, and showed areas of fibrinoid necrosis, suggestive of mechanical abrasion and increased pressure.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2686040/

Record 1554

pmcid: PMC1971048

pmid: 17650327

doi: 10.1186/1745-0179-3-12

title: Low dose quetiapine induced galactorrhoea: a case report.

authors

- Gupta M

journal: Clinical practice and epidemiology in mental health : CP & EMH

journalAbbr: Clin Pract Epidemiol Ment Health

year: 2007

pubdate: 2007 Jul 24

volume: 3

issue:

pages: 12

abstract: BACKGROUND: Quetiapine causes less prolactin elevation and/or galactorrhoea than other atypical antipsychotics. CASE PRESENTATION: Ms AB had galactorrhoea and raised prolactin levels at only 100 mg of quetiapine daily. CONCLUSION: Low dose quetiapine can also cause galactorrhoea.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1971048/

Record 1555

pmcid: PMC2360003

pmid: 17242703

doi: 10.1038/sj.bjc.6603573

title: Mutation analysis of aryl hydrocarbon receptor interacting protein (AIP) gene in colorectal, breast, and prostate cancers.

authors

- Georgitsi M
- Karhu A
- Winqvist R
- Visakorpi T
- Waltering K
- Vahteristo P
- Launonen V
- Aaltonen LA

journal: British journal of cancer

journalAbbr: Br J Cancer

year: 2007

pubdate: 2007 Jan 29

volume: 96

issue: 2

pages: 352-6

abstract: Germline mutations in the aryl hydrocarbon receptor interacting protein (AIP) gene were recently identified in individuals with pituitary adenoma predisposition (PAP). These patients have prolactin (PRL) or growth hormone (GH) oversecreting pituitary adenomas, the latter exhibiting acromegaly or gigantism. Loss-of-heterozygosity (LOH) analysis revealed that AIP is lost in PAP tumours, suggesting that it acts as a tumour-suppressor gene. Aryl hydrocarbon receptor interacting protein is involved in several pathways, but it is best characterised as a cytoplasmic partner of the aryl hydrocarbon receptor (AHR). To examine the possible role of AIP in the genesis of common cancers, we performed somatic mutation screening in a series of 373 colorectal cancers (CRCs), 82 breast cancers, and 44 prostate tumour samples. A missense R16H (47G>A) change was identified in two CRC samples, as well as in the respective normal tissues, but was absent in 209 healthy controls. The remaining findings were silent, previously unreported, changes of the coding, non-coding, or untranslated regions of AIP. These results suggest that somatic AIP mutations are not common in CRC, breast, and prostate cancers.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2360003/

Record 1556

pmcid: PMC1936299

pmid: 18360626

doi: 10.2147/tcrm.2007.3.1.181

title: Outcomes and quality of life issues in the pharmacological management of benign prostatic hyperplasia (BPH).

authors

- Cambio AJ
- Evans CP

journal: Therapeutics and clinical risk management

journalAbbr: Ther Clin Risk Manag

year: 2007

pubdate: 2007 Mar

volume: 3

issue: 1

pages: 181-96

abstract: BACKGROUND: Benign prostatic hyperplasia (BPH) is a common disease of the aging male population. BPH treatment includes a variety of pharmacological and surgical interventions. The goal of this paper is to review the natural history of BPH, outcomes of pharmacological management, effects on quality of life (QoL), future pharmacotherapies, and associated patient-focused perspectives. MATERIALS AND METHODS: Medline searches for the keywords benign prostatic hyperplasia, BPH, alpha blockers, 5 alpha-reductase, and quality of life were performed. Relevant literature was reviewed and analyzed. RESULTS: Alpha blockers, 5 alpha-reductase inhibitors, and phytotherapy are the three categories of pharmaceutical interventions currently available for BPH. Various clinical trials have shown that alpha blockers and 5 alpha-reductase inhibitors are safe, efficacious, and improve QoL in patients with BPH. The evidence for phytotherapeutics is not as convincing. The current armamentarium of pharmaceutical interventions are encompassed in these three classes of medications. New pharmacotherapies based on novel mechanisms are on the horizon. CONCLUSION: There are a variety of safe and efficacious medical therapies available for the management of BPH and it is important for the practicing physician to have an understanding of these pharmacotherapies and their potential impact on the patient. There is not enough evidence to make a recommendation regarding phytotherapy use. New classes of drugs for BPH will likely find their way into routine use.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1936299/

Record 1557

pmcid: PMC2376090

pmid: 18473017

doi:

title: Pharmacological causes of hyperprolactinemia.

authors

- Torre DL
- Falorni A

journal: Therapeutics and clinical risk management

journalAbbr: Ther Clin Risk Manag

year: 2007

pubdate: 2007 Oct

volume: 3

issue: 5

pages: 929-51

abstract: Hyperprolactinemia is a common endocrinological disorder that may be caused by several physiological and pathological conditions. Several drugs may determine a significant increase in prolactin serum concentration that is frequently associated with symptoms. The so-called typical antipsychotics are frequently responsible for drug-related hyperprolactinemia. Risperidone is one of the atypical neuroleptics most likely to induce hyperprolactinemia, while other atypical drugs are unfrequenlty and only transiently associated with increase of prolactin levels. Women are more sensitive than men to the hyperprolactinemic effect of antipsychotics. Classical and risperidone-induced hyperprolactinemia may be revert when a gradual antipsychotic drug discontinuation is combined with olanzapine or clozapine initiation. Antidepressant drugs with serotoninergic activity, including selective serotonin reuptake inhibitors (SSRI), monoamine oxidase inhibitors (MAO-I) and some tricyclics, can cause hyperprolactinemia. A long list of other compounds may determine an increase in prolactin levels, including prokinetics, opiates, estrogens, anti-androgens, anti-hypertensive drugs, H2-receptor antagonists, anti-convulsivants and cholinomimetics. Finally, hyperprolactinemia has also been documented during conditioning and after autologous blood stem-cell transplantation and during chemotherapy, even though disturbances of prolactin seem to occur less frequently than impairments of the hypothalamus-pituitary-gonad/thyroid axis after intensive treatment and blood marrow transplantation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2376090/

Record 1558

pmcid: PMC4895095

pmid: 27303482

doi: 10.2484/rcr.v2i3.96

title: Recurrent Bilateral Giant Fibroadenomas of the Breasts.

authors

- Morris A
- Shaffer K

journal: Radiology case reports

journalAbbr: Radiol Case Rep

year: 2007

pubdate: 2007

volume: 2

issue: 3

pages: 96

abstract: We present a case of an 18-year-old woman with recurrent bilateral giant fibroadenomas that were evaluated by mammography and sonography with color Doppler. Imaging revealed solid lobulated masses with significant internal vascularity occupying most of each breast; this evaluation suggested a differential diagnosis of giant fibroadenoma versus phyllodes tumor. The unusual clinical story of recurrent bilateral lesions as reported by the patient, coupled with the findings on visualization of these lesions by mammography and ultrasound with color Doppler, led to the clinical decision to forego biopsy in favor of immediate bilateral surgical enucleation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4895095/

Record 1559

pmcid: PMC2598213

pmid: 18055907

doi: 10.1136/jme.2006.017095

title: Teaching medical students about fair distribution of healthcare resources.

authors

- Leget C
- Hoedemaekers R

journal: Journal of medical ethics

journalAbbr: J Med Ethics

year: 2007

pubdate: 2007 Dec

volume: 33

issue: 12

pages: 737-41

abstract: Healthcare package decisions are complex. Different judgements about effectiveness, cost-effectiveness and disease burden influence the decision-making process. Moreover, different concepts of justice generate different ideas about fair distribution of healthcare resources. This paper presents a decision model that is used in medical school in order to familiarise medical students with the different concepts of justice and the ethical dimension of making concrete choices. The model is based on the four-stage decision model developed in the Netherlands by the Dunning Committee and the discussion that followed its presentation in 1991. Having to deal with 10 medical services, students working with the model learn to discern and integrate four different ideas of distributive justice that are integrated in a flow chart: libertarian, communitarian, egalitarian and utilitarian.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2598213/

Record 1560

pmcid: PMC2151151

pmid:

doi: 10.1136/bmj.38994.462801.DE

title: The computer.

authors

- Isaacs D
- Isaacs S
- Fitzgerald D

journal: BMJ : British Medical Journal

journalAbbr: BMJ

year: 2007

pubdate: 2007 Dec 22

volume: 335

issue: 7633

pages: 1304

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2151151/

Record 1561

pmcid: PMC1475602

pmid: 16600050

doi: 10.1186/1742-6405-3-10

title: Antiretroviral therapy at a district hospital in Ethiopia prevents death and tuberculosis in a cohort of HIV patients.

authors

- Jerene D
- Naess A
- Lindtjørn B

journal: AIDS research and therapy

journalAbbr: AIDS Res Ther

year: 2006

pubdate: 2006 Apr 7

volume: 3

issue:

pages: 10

abstract: BACKGROUND: Although highly active antiretroviral therapy (HAART) reduces mortality in the developed world, it remains undocumented in resource-poor settings. We assessed the effect of HAART on patient mortality and tuberculosis incidence rate under routine clinical care conditions in Ethiopia. The objective of this study was to assess the effect of HAART on patient mortality and tuberculosis incidence rate under routine clinical care conditions in a resource-limited setting in south Ethiopia. Starting in January 2003, we followed all consecutive adult HIV infected patients who visited the HIV clinic. Since August 2003, we treated patients with HAART. Only basic laboratory services were available. RESULTS: We followed 185 patients in the pre-HAART cohort and 180 patients in the HAART cohort. The mortality rate was 15.4 per 100 person-years of observation (PYO) in the HAART group and tuberculosis incidence rate was 3.7 per 100 PYO. In the pre-HAART group, the mortality rate was 58.1 per 100 PYO and the tuberculosis incidence rate was 11.1 per 100 PYO. HAART resulted in a 65% decline in mortality (adjusted hazard ratio [95%CI] = 0.35 [0.19-0.63]; P < 0.001). Tuberculosis incidence rate was lower in the HAART group (adjusted hazard ratio [95%CI] = 0.11 [0.03-0.48]; P < 0.01). Most of the deaths occurred during the first three months of treatment. CONCLUSION: HAART improved survival and decreased tuberculosis incidence to a level similar to that achieved in the developed countries during the early years of HAART. However, both the mortality and the tuberculosis incidence rate were much higher in terms of absolute figures in this resource-limited setting. Attention should be paid to the early weeks of treatment when mortality is high. The high tuberculosis incidence rate, when coupled with the improved survival, may lead to increased tuberculosis transmission. This highlights the need for strengthening tuberculosis prevention efforts with the scale-up of treatment programmes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1475602/

Record 1562

pmcid: PMC1482340

pmid: 16793811

doi: 10.1136/bmj.332.7556.1489

title: Antiretroviral treatment of HIV infected adults.

authors

- Deeks SG

journal: BMJ (Clinical research ed.)

journalAbbr: BMJ

year: 2006

pubdate: 2006 Jun 24

volume: 332

issue: 7556

pages: 1489

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1482340/

Record 1563

pmcid: PMC2686055

pmid: 19554139

doi: 10.1177/229255030601400403

title: Autoinflation of saline-filled inflatable breast implants.

authors

- Peters W

journal: The Canadian journal of plastic surgery = Journal canadien de chirurgie plastique

journalAbbr: Can J Plast Surg

year: 2006

pubdate: 2006 Winter

volume: 14

issue: 4

pages: 219-26

abstract: Spontaneous autoinflation of saline-filled breast implants is a rare phenomenon; only 20 cases have been reported in the world literature. Over the past seven years, three patients have presented with significant unilateral autoinflation of their smooth, single-lumen, round, saline-filled implants. This developed at various times: progressively over 23 years with a Simaplast implant; between the ninth and 10th year after augmentation with a leaflet valve implant; and slowly over the first four years with a leaflet valve implant. The etiology of the autoinflation was shown to be different for the two types of implants. The Simaplast implant had likely been injected with a hypertonic filling solution - one that was twice as concentrated as 'normal saline'. This would have created an osmotic gradient, which would have facilitated autoexpansion by diffusion. The implant solution remained clear and transparent. In addition, there were no detectable levels of glucose, uric acid or albumin in the fluid. By contrast, auto-inflation of the leaflet valve implants likely resulted from mechanical alterations of the valve mechanism. This type of implant is known to have a high deflation rate, with frequent partial deflations. It is interesting that one of the patients with the leaflet valve implants presented with an autoexpansion on one side and a partial deflation on the other side. Both implants were from the same lot number. The same mechanism that caused partial deflations may have also allowed fluid from the implant pocket to pass through the valve into the lumen of these implants. This could allow glucose, protein and cellular elements to enter into the lumen (these would not pass through an intact elastomeric shell) which would create an osmotic gradient, allowing water to enter the elastomeric shell by diffusion. The fluid in these leaflet valve implants was brownish yellow, very viscous and turbid. It contained elevated levels of glucose and uric acid which would not have passed through the elastomeric shell. Over the past 10 years, four different theories have been proposed in an attempt to explain the etiology of autoinflation. However, the findings of the present study indicate that there are only two mechanisms - a hypertonic filling solution and alterations of the valve mechanism.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2686055/

Record 1564

pmcid: PMC7158182

pmid:

doi: 10.1016/B0-72-160537-0/50012-8

title: Focal Bacterial Infections.

authors

- Overturf GD

journal: Infectious Diseases of the Fetus and Newborn Infant

journalAbbr: Infectious Diseases of the Fetus and Newborn Infant

year: 2006

pubdate: 2006

volume:

issue:

pages: 347-84

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7158182/

Record 1565

pmcid: PMC8109503

pmid: 17170615

doi: 10.1111/j.1524-6175.2006.06107.x

title: Primary aldosteronism: diagnosis and treatment.

authors

- Pimenta E
- Calhoun DA

journal: Journal of clinical hypertension (Greenwich, Conn.)

journalAbbr: J Clin Hypertens (Greenwich)

year: 2006

pubdate: 2006 Dec

volume: 8

issue: 12

pages: 887-93

abstract: Recent studies have indicated a higher prevalence of primary aldosteronism (PA) than reported historically. Aldosterone excess induces sodium and fluid retention with consequential increases in blood pressure. Patients with PA are at an increased risk of developing left ventricular hypertrophy, chronic kidney disease, and endothelial dysfunction. Measurement of the plasma aldosterone/plasma renin activity ratio is an effective screening test for PA. The majority of patients with PA do not have a discernable aldosterone-producing adenoma (APA), and the aldosterone excess is considered idiopathic in etiology and/or attributed to adrenal hyperplasia. Treatment of PA includes medical therapy with mineralocorticoid receptor antagonists and adrenalectomy for patients with a unilateral APA. A reasonable treatment strategy is to attempt medical therapy in all patients with a high plasma aldosterone/PRA ratio and reserve the extensive workup needed to identify an APA for those patients whose hypertension or hypokalemia cannot be controlled medically.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8109503/

Record 1566

pmcid: PMC2699636

pmid: 18046919

doi: 10.2147/ciia.2006.1.4.425

title: Role of 5 alpha-reductase inhibitors in the management of prostate cancer.

authors

- Hudak SJ
- Hernandez J
- Thompson IM

journal: Clinical interventions in aging

journalAbbr: Clin Interv Aging

year: 2006

pubdate: 2006

volume: 1

issue: 4

pages: 425-31

abstract: Prostate cancer is one of the most complex and enigmatic oncologic problems in medicine. It is highly prevalent, particularly in elderly males. Unfortunately, its generally protracted and variable clinical course and high association with treatment-related morbidity raise serious questions about the ideal treatment strategy for the individual patient. 5 alpha-reductase (5AR) inhibitors have a dramatic effect on benign prostatic disease with low toxicity. Thus, there is much interest in the potential role of 5AR inhibitors in the prevention and treatment of prostate cancer. Finasteride is the only agent that has been shown in a randomized clinical trial to decrease the risk of prostate cancer with a reduction of almost 25%. Additionally, a recent analysis of the Prostate Cancer Prevention Trial (PCPT) has found that finasteride improves the performance characteristics of prostate-specific antigen (PSA) blood test as a screening tool for prostate cancer, for both cancer detection as well as for detection of high risk disease. Finally, 5AR inhibitors have been studied as a component of multimodal therapy for all stages of prostate cancer, with the goal of improving oncologic outcomes while avoiding the toxicity of medical and surgical castration.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2699636/

Record 1567

pmcid: PMC1432188

pmid: 16601045

doi: 10.1136/bmj.332.7545.837

title: Testicular tumour presenting as gynaecomastia.

authors

- Harris M
- Rizvi S
- Hindmarsh J
- Bryan R

journal: BMJ (Clinical research ed.)

journalAbbr: BMJ

year: 2006

pubdate: 2006 Apr 8

volume: 332

issue: 7545

pages: 837

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1432188/

Record 1568

pmcid: PMC6654241

pmid: 16649723

doi: 10.1002/clc.4960290405

title: Use and side-effect profile of spironolactone in a private cardiologist's practice.

authors

- Williams EM
- Katholi RE
- Karambelas MR

journal: Clinical cardiology

journalAbbr: Clin Cardiol

year: 2006

pubdate: 2006 Apr

volume: 29

issue: 4

pages: 149-53

abstract: BACKGROUND: The beneficial effects of spironolactone on the treatment of cardiovascular diseases are well known, but translating these benefits into private practice can be difficult because of the drug's side-effect profile. HYPOTHESIS: When patients are monitored over the long term, spironolactone can be used safely with an acceptable side-effect profile. METHODS: We retrospectively studied 762 patients taking spironolactone over a 7-year period in a cardiologist's referral-based practice and monitored them for side effects from the medication. RESULTS: Data were available on 762 patients. The average age of our patients when started on the medication was 67.2 +/- 0.5 years. Of these, 585 (76.8%) patients were treated for heart failure and 155 (20.3%) for hypertension. An average dose of 38.4 +/- 1.4 mg of spironolactone was used for treatment of all conditions. Of the 762 patients, 81 (10.6%) experienced side effects while using the medication; 40 had hyperkalemia (5.3%), 14 had gynecomastia (1.8%), and 15 had gastritis (2%). Of the patients with hyperkalemia, average creatinine clearance decreased from 64.6 +/- 5.8 ml/min at therapy start to 50.3 +/- 5.5 ml/min at the time of onset of side effects. CONCLUSION: Spironolactone can be used with an acceptable side-effect profile as long as patients are monitored long-term while receiving the medication.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6654241/

Record 1569

pmcid: PMC2361224

pmid: 16523201

doi: 10.1038/sj.bjc.6603017

title: Use of antidepressant medications in relation to the incidence of breast cancer.

authors

- Fulton-Kehoe D
- Rossing MA
- Rutter C
- Mandelson MT
- Weiss NS

journal: British journal of cancer

journalAbbr: Br J Cancer

year: 2006

pubdate: 2006 Apr 10

volume: 94

issue: 7

pages: 1071-8

abstract: Although associations have been reported between antidepressant use and risk of breast cancer, the findings have been inconsistent. We conducted a population-based case-control study among women enrolled in Group Health Cooperative (GHC), a health maintenance organization in Washington State. Women with a first primary breast cancer diagnosed between 1990 and 2001 were identified (N = 2904) and five controls were selected for each case (N = 14396). Information on antidepressant use was ascertained through the GHC pharmacy database and on breast cancer risk factors and screening mammograms from GHC records. Prior to one year before diagnosis of breast cancer, about 20% of cases and controls had used tricyclic antidepressants (adjusted odds ratio = 1.06, 95% CI 0.94-1.19) and 6% of each group had used selective serotonin reuptake inhibitors (OR = 0.98, 95% CI 0.80-1.18). There also were no differences between cases and controls with regard to the number of prescriptions filled or the timing of use. Taken as a whole, the results from this and other studies to date do not indicate an altered risk of breast cancer associated with the use of antidepressants overall, by class, or for individual antidepressants.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2361224/

Record 1570

pmcid: PMC1064128

pmid: 15743498

doi: 10.1186/bcr974

title: Breast implants following mastectomy in women with early-stage breast cancer: prevalence and impact on survival.

authors

- Le GM
- O'Malley CD
- Glaser SL
- Lynch CF
- Stanford JL
- Keegan TH
- West DW

journal: Breast cancer research : BCR

journalAbbr: Breast Cancer Res

year: 2005

pubdate: 2005

volume: 7

issue: 2

pages: R184-93

abstract: BACKGROUND: Few studies have examined the effect of breast implants after mastectomy on long-term survival in breast cancer patients, despite growing public health concern over potential long-term adverse health effects. METHODS: We analyzed data from the Surveillance, Epidemiology and End Results Breast Implant Surveillance Study conducted in San Francisco-Oakland, in Seattle-Puget Sound, and in Iowa. This population-based, retrospective cohort included women younger than 65 years when diagnosed with early or unstaged first primary breast cancer between 1983 and 1989, treated with mastectomy. The women were followed for a median of 12.4 years (n = 4968). Breast implant usage was validated by medical record review. Cox proportional hazards models were used to estimate hazard rate ratios for survival time until death due to breast cancer or other causes for women with and without breast implants, adjusted for relevant patient and tumor characteristics. RESULTS: Twenty percent of cases received postmastectomy breast implants, with silicone gel-filled implants comprising the most common type. Patients with implants were younger and more likely to have in situ disease than patients not receiving implants. Risks of breast cancer mortality (hazard ratio, 0.54; 95% confidence interval, 0.43-0.67) and nonbreast cancer mortality (hazard ratio, 0.59; 95% confidence interval, 0.41-0.85) were lower in patients with implants than in those patients without implants, following adjustment for age and year of diagnosis, race/ethnicity, stage, tumor grade, histology, and radiation therapy. Implant type did not appear to influence long-term survival. CONCLUSIONS: In a large, population-representative sample, breast implants following mastectomy do not appear to confer any survival disadvantage following early-stage breast cancer in women younger than 65 years old.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1064128/

Record 1571

pmcid: PMC2782171

pmid: 16100467

doi: 10.3346/jkms.2005.20.4.683

title: Cutaneous metastasis of choriocarcinoma: a case report.

authors

- Park SG
- Chang JY
- Kim SH
- Bang D

journal: Journal of Korean medical science

journalAbbr: J Korean Med Sci

year: 2005

pubdate: 2005 Aug

volume: 20

issue: 4

pages: 683-6

abstract: Choriocarcinoma is one of the malignant tumors of trophoblastic cells characterized by the secretion of human chorionic gonadotrophin (hCG) (1-3). Cutaneous metastasis is a rare presentation of choriocarcinoma but a poor prognostic sign because it is associated only with widespread disease (3-5). A 52-yr-old female complaining of dyspnea for 2 months, presented with fingertip sized erythematous nodules on the left side of the neck and the right side of the upper back of 1 month duraton. She has suffered from Behcet's disease since

1999. Microscopic examination of a nodule of upper back demonstrated biphasic pattern of cytotrophoblasts and hCG-positive syncytiotrophoblasts, and the typical histologic features of choriocarcinoma. She was referred to the gynecological oncology department. After 17 cycles of combination chemotherapy, the serum hCG level has fallen from 700,000 to under 2.0 mIU/mL and the skin lesions have almost disappeared. However, after 3 months, total abdominal hysterectomy and bilateral salpingo-oophorectomy were performed due to elevated serum hCG level (4,447.7 mIU/mL), and she is scheduled to receive post-operative adjuvant chemotherapy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2782171/

Record 1572

pmcid: PMC4615444

pmid: 16015691

doi: 10.3748/wjg.v11.i27.4210

title: Effect of itopride, a new prokinetic, in patients with mild GERD: a pilot study.

authors

- Kim YS
- Kim TH
- Choi CS
- Shon YW
- Kim SW
- Seo GS
- Nah YH
- Choi MG
- Choi SC

journal: World journal of gastroenterology

journalAbbr: World J Gastroenterol

year: 2005

pubdate: 2005 Jul 21

volume: 11

issue: 27

pages: 4210-4

abstract: AIM: Itopride is a newly developed prokinetic agent, which enhances gastric motility through both antidopaminergic and anti-acetylcholinesterasic actions. The importance of esophageal motor dysfunction in the pathogenesis of gastro-esophageal reflux disease (GERD) makes it interesting to examine the effect of itopride on esophageal acid exposure. METHODS: The effect of itopride on esophageal acid reflux variables for 24 h was studied in 26 patients with GERD symptoms, pre-entry total acid exposure time (pH<4) of more than 5% and mild esophagitis (Savary-Miller grades I, II) proven by endoscopy. Ambulatory 24-h pH-metry and symptom assessment were performed after treatments with 150 or 300 mg itopride thrice a day (t.i.d.) for 30 d in random order, using an open label method. For evaluating the safety of itopride, blood biochemical laboratory test was performed and the serum prolactin level was also examined before and after treatment. RESULTS: Total symptom score was significantly decreased after treatment in 150- or 300-mg group. Itopride 300 mg was significantly effective than 150 mg on decreasing the total per cent time with pH<4, total time with pH<4 and DeMeester score. No serious adverse effects were observed with administration of itopride in both groups. CONCLUSION: Itopride 100 mg t.i.d. is effective on decreasing pathologic reflux in patient with GERD and therefore it has the potential to be effective in the treatment of this disease.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4615444/

Record 1573

pmcid: PMC3410128

pmid: 23012082

doi:

title: Giant juvenile fibroadenoma: experience from a university hospital.

authors

- Abdelhadi MS

journal: Journal of family & community medicine

journalAbbr: J Family Community Med

year: 2005

pubdate: 2005 May

volume: 12

issue: 2

pages: 91-5

abstract: OBJECTIVE: Fibroadenoma is one of the commonest benign breast lesions in our outpatient clinics. Giant Juvenile fibroadenoma (GJF) characterized with their alarming rapid growth and gross disfigurement is less frequently identified. MATERIALS AND METHODS: A 14-year review (1990-2004) of all fibroadenomas presented to the Outpatient Department was undertaken. Demographic data, duration of symptoms, size at presentation, the use of radiological, cytological and histo-pathological modalities, surgical excision and follow up were all noted. Eight-Hundred-Sixty-Four cases were diagnosed as fibroadenoma by both clinical and radiological examinations and confirmed by FNAC. Patients with fibroadenomas <2 cm in size were followed up regularly in the out patients department, while those >2 cm underwent surgical excision. GJF were defined as those with >5 cm in diameter. RESULTS: The total number of excised fibroadenomas was 202 (23%), while the remaining662 (77%) fell into the follow up category. GJF were diagnosed in 9 patients accounting for 4.5% of all excised fibroadenomas. Age ranged between 14-23 years. CONCLUSION: However benign these lesions may appear, in view of the history of a sudden rapid breast enlargement as demonstrated in nearly all the clinical presentations, surgical excision remains the mainstay of treatment of such lesions in order to allow the previously compressed normal surrounding breast tissue to expand and retain its normal function and cosmetic appearance. The use of radiological modalities such as ultrasound and MRI may aid the diagnosis, limiting mammography to the older age group.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3410128/

Record 1574

pmcid: PMC8109438

pmid: 15860966

doi: 10.1111/j.1524-6175.2005.04105.x

title: Gynecomastia and hypertension.

authors

- Prisant LM
- Chin E

journal: Journal of clinical hypertension (Greenwich, Conn.)

journalAbbr: J Clin Hypertens (Greenwich)

year: 2005

pubdate: 2005 Apr

volume: 7

issue: 4

pages: 245-8

abstract: Palpable dense and mobile subareolar tissue in the male breast defines the presence of gynecomastia. For the hypertension specialist, breast enlargement in men provides a clue to a secondary cause of hypertension or an adverse antihypertensive drug reaction. Hyperthyroidism, chronic renal failure, adrenal hyperplasia or tumors, amphetamine, cyclosporine, and anabolic steroids are secondary causes of hypertension associated with gynecomastia. Reserpine, methyldopa, and spironolactone are older drugs associated with gynecomastia; however, calcium antagonists (more commonly), angiotensin-converting enzyme inhibitors, and alpha1 blockers may also be associated with this finding. Treatment is directed to removal of the underlying cause.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC8109438/

Record 1575

pmcid: PMC3004714

pmid: 21179629

doi:

title: Normalization of risperidone-induced hyperprolactinemia with the addition of aripiprazole.

authors

- Shores LE

journal: Psychiatry (Edgmont (Pa. : Township))

journalAbbr: Psychiatry (Edgmont)

year: 2005

pubdate: 2005 Mar

volume: 2

issue: 3

pages: 42-5

abstract: The objective of this study was to monitor metabolic changes, including hyperprolactinemia, in adolescents medicated with atypical antipsychotics, especially when polypharmacy is involved. This study specifically followed risperidone-induced hyperprolactinemia in adolescents (14 male patients and 2 female patients) after aripiprazole was added to begin transitioning to another atypical antipsychotic. No other changes were made in the medication regimen. Risperidone was continued at the previous dose during the transition. This sample consists of 16 adolescents, 13 to 18 years old, treated in long-term residential care for behavior problems. Diagnoses included bipolar disorder, schizoaffective disorder, psychotic depression, and post-traumatic stress disorder. In retrospective chart review, all 16 of the adolescents (100%) experienced complete normalization of prolactin levels. Thirteen of the adolescents had normal prolactin levels within two weeks and the other three normalized within three months of the addition of aripiprazole while still on risperidone and other previous medications at the original doses. The authors concluded the addition of aripiprazole rapidly normalizes risperidone-induced hyperprolactinemia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3004714/

Record 1576

pmcid: PMC2823034

pmid: 15744799

doi: 10.3349/ymj.2005.46.1.1

title: Unilateral breast edema: spectrum of etiologies and imaging appearances.

authors

- Kwak JY
- Kim EK
- Chung SY
- You JK
- Oh KK
- Lee YH
- Kwon TH
- Jung HK

journal: Yonsei medical journal

journalAbbr: Yonsei Med J

year: 2005

pubdate: 2005 Feb 28

volume: 46

issue: 1

pages: 1-7

abstract: Breast edema is defined as a mammographic pattern of skin thickening, increased parenchymal density, and interstitial marking. It can be caused by benign or malignant diseases, as a result of a tumor in the dermal lymphatics of the breast, lymphatic congestion caused by breast, lymphatic drainage obstruction, or by congestive heart failure. Here we describe several conditions, that cause unilateral breast edema with the aim of familiarizing radiologists with these disease entities.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2823034/

Record 1577

pmcid: PMC1964013

pmid: 15032220

doi:

title: A simple, safe and effective method for laparoscopic port closure.

authors

- Ng WT

journal: Annals of the Royal College of Surgeons of England

journalAbbr: Ann R Coll Surg Engl

year: 2004

pubdate: 2004 Jan

volume: 86

issue: 1

pages: 71-2; author reply 73

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1964013/

Record 1578

pmcid: PMC1964034

pmid: 15032216

doi:

title: Breast augmentation should be on the NHS: a discussion of the ethics of rationing.

authors

- Henderson J

journal: Annals of the Royal College of Surgeons of England

journalAbbr: Ann R Coll Surg Engl

year: 2004

pubdate: 2004 Jan

volume: 86

issue: 1

pages: 69-70

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1964034/

Record 1579

pmcid: PMC1964035

pmid: 15032215

doi:

title: Breast augmentation should be on the NHS: a discussion of the ethics of rationing.

authors

- Davis BJ
- Jindal M
- Kisku W

journal: Annals of the Royal College of Surgeons of England

journalAbbr: Ann R Coll Surg Engl

year: 2004

pubdate: 2004 Jan

volume: 86

issue: 1

pages: 69

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1964035/

Record 1580

pmcid: PMC1472914

pmid: 16985923

doi:

title: Comparison of clinical trials with finasteride and dutasteride.

authors

- Nickel JC

  journal: Reviews in urology

  journalAbbr: Rev Urol

  year: 2004

  pubdate: 2004

  volume: 6 Suppl 9

  issue: Suppl 9

  pages: S31-9

  abstract: Finasteride selectively inhibits the Type 2 isoenzyme of 5alpha-reductase (5AR) (the enzyme responsible for converting testosterone to dihydrotestosterone [DHT]) whereas dutasteride inhibits both Type 1 and Type 2 5AR. General conclusions regarding the differences and similarities of these 2 agents, in terms of pharmacologic effect, safety, and efficacy, can be drawn from evaluation of short-term comparative trials and similar but non-comparative long-term trials. Dutasteride therapy reduces serum DHT significantly more than does finasteride. In men with benign prostatic hyperplasia (BPH), treatment with either agent results in similar prostate gland volume reduction, flow rate and symptom improvement, and similar reductions in long-term risk of BPH development in terms of symptom progression and acute urinary retention (AUR) and BPH-related surgery. There does not appear to be any clinically significant difference between the adverse event profiles of dutasteride and finasteride. Although weak evidence suggests a difference in the onset of clinical benefit, the available non-comparative trial data do not confirm this finding. Patients with symptomatic BPH who receive dutasteride or finasteride, either as monotherapy or combination therapy with alpha-blockers, can expect to experience significant prostate gland size reduction, improved symptoms, and reduced risk of progression in terms of long-term adverse outcomes.

  url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1472914/

Record 1581

pmcid: PMC1964016

pmid: 15032218

doi:

title: Compartment syndrome following pelvic surgery in the lithotomy position.

authors

- Thomson WH

  journal: Annals of the Royal College of Surgeons of England

  journalAbbr: Ann R Coll Surg Engl

  year: 2004

  pubdate: 2004 Jan

  volume: 86

  issue: 1

  pages: 70

  abstract:

  url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1964016/

Record 1582

pmcid: PMC1964009

pmid: 15032217

doi:

title: Compartment syndrome following pelvic surgery in the lithotomy position.

authors

- Lagattolla NR
- Bond J

journal: Annals of the Royal College of Surgeons of England

journalAbbr: Ann R Coll Surg Engl

year: 2004

pubdate: 2004 Jan

volume: 86

issue: 1

pages: 70

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1964009/

Record 1583

pmcid: PMC1964020

pmid: 15032221

doi:

title: Finding the best from the rest: evaluation of the quality of patient information on the Internet.

authors

- Knight T

journal: Annals of the Royal College of Surgeons of England

journalAbbr: Ann R Coll Surg Engl

year: 2004

pubdate: 2004 Jan

volume: 86

issue: 1

pages: 73

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1964020/

Record 1584

pmcid: PMC6147906

pmid: 15323281

doi: 10.5144/0256-4947.2004.145b

title: Precocious puberty: an unusual presentation of juvenile hypothyroidism.

authors

- Vakili R

journal: Annals of Saudi medicine

journalAbbr: Ann Saudi Med

year: 2004

pubdate: 2004 Mar-Apr

volume: 24

issue: 2

pages: 145-7

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6147906/

Record 1585

pmcid: PMC1964021

pmid: 15032219

doi:

title: Problem-based learning for surgical trainees.

authors

- Singh Ranger G

journal: Annals of the Royal College of Surgeons of England

journalAbbr: Ann R Coll Surg Engl

year: 2004

pubdate: 2004 Jan

volume: 86

issue: 1

pages: 70-1; author reply 71

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1964021/

Record 1586

pmcid: PMC1472918

pmid: 16985926

doi:

title: Role of Primary Care Clinicians in the Diagnosis and Treatment of LUTS and BPH.

authors

- Kuritzky L

journal: Reviews in urology

journalAbbr: Rev Urol

year: 2004

pubdate: 2004

volume: 6 Suppl 9

issue: Suppl 9

pages: S53-9

abstract: Primary care clinicians are often responsible for the treatment of lower urinary tract symptoms (LUTS) and benign prostatic hyperplasia (BPH). The combination of the compelling epidemiologic presence of BPH, compromised quality of life due to LUTS, and the availability of highly effective oral therapeutic agents, offers an opportunity for substantial clinical impact in the primary care setting. Evolving management pathways include utilization of both symptom-modifying treatment and disease-modifying treatment. Alpha blockers are excellent to provide symptomatic treatment, but do not alter long-term disease progression. Alpha reductase inhibitors provide therapy by reducing the need for surgical intervention and the incidence of acute urinary retention. The combination of alpha blockers and alpha reductase inhibitors will be best for some patients, typically those with large prostate glands in whom disease progression is most likely. Primary care clinicians will want to become more familiar with recent clinical trails that are shaping emerging therapeutic practice.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1472918/

Record 1587

pmcid: PMC2409882

pmid: 15280915

doi: 10.1038/sj.bjc.6602071

title: The utility of a multimedia education program for prostate cancer patients: a formative evaluation.

authors

- Flynn D
- van Schaik P
- van Wersch A
- Ahmed T
- Chadwick D

journal: British journal of cancer

journalAbbr: Br J Cancer

year: 2004

pubdate: 2004 Aug 31

volume: 91

issue: 5

pages: 855-60

abstract: A multimedia program (MMP) was developed to educate patients with prostate cancer about their disease. A within-subjects design was used to investigate the changes in levels of cancer-related knowledge, psychosocial functioning, treatment decision-making role and information needs immediately after browsing the MMP. The participants were 67 men recently diagnosed with prostate cancer. Psychosocial functioning was assessed with 20 items describing common emotional states and coping strategies employed by cancer patients. Treatment decision-making role was assessed with the Control Preference Scale. A principle component analysis of the 20 psychosocial items yielded three components: distress, positive approach and nonacceptance. After browsing the MMP significant increases in knowledge and reductions in distress were reported. Marital status was significantly associated with knowledge gain. Married men and those attending the study session with their spouse displayed a significant shift towards a more active role in treatment decisions. The majority of information needs were fulfilled by the MMP; however, information related to the likelihood of a cure, treatment side effects, coping strategies and aetiology were not completely satisfied by the MMP. Implications of the findings and suggestions for future work on the design and evaluation of the MMP are discussed.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2409882/

Record 1588

pmcid: PMC1964024

pmid: 15032222

doi:

title: Wroblewski wedge augmentation for recurrent posterior dislocation of the Charnley total hip replacement.

authors

- Atkinson JJ
- Webb MR
- Lovell ME

journal: Annals of the Royal College of Surgeons of England

journalAbbr: Ann R Coll Surg Engl

year: 2004

pubdate: 2004 Jan

volume: 86

issue: 1

pages: 74; author reply 74

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1964024/

Record 1589

pmcid: PMC1241552

pmid: 12826475

doi: 10.1289/ehp.6023

title: A critical review of methods for comparing estrogenic activity of endogenous and exogenous chemicals in human milk and infant formula.

authors

- Borgert CJ
- LaKind JS
- Witorsch RJ

journal: Environmental health perspectives

journalAbbr: Environ Health Perspect

year: 2003

pubdate: 2003 Jun

volume: 111

issue: 8

pages: 1020-36

abstract: The two primary sources of nutrition for infants are human milk and infant formula. Both contain an array of endogenous and exogenous chemicals that may act through many separate hormonal mechanisms. The safety of infant nutrition sources has been questioned based on the possibility that exogenous chemicals may exert adverse effects on nursing or formula-fed infants through estrogen-mediated mechanisms. In response to these and other concerns, the National Research Council recommended assessing the estrogenic potency of natural and anthropogenic hormonally active agents. Furthermore, the Endocrine Disruptor Screening and Testing Advisory Committee of the U.S. Environmental Protection Agency specifically

recommended testing chemicals present in human milk as a representative mixture to which large segments of the population are exposed. To date, no clinical or epidemiologic evidence demonstrates that levels of chemicals currently found in human milk or infant formulas cause adverse effects in infants. Nonetheless, the question is sufficiently important to warrant a consideration of how best to evaluate potential estrogenic risks. We reviewed the types of data available for measuring estrogenic potency as well as methods for estimating health risks from mixtures of chemicals in infant nutrition sources that act via estrogenic mechanisms. We conclude that the science is insufficiently developed at this time to allow a credible assessment of health risks to infants based on estimates of estrogenic potency or on an understanding of toxicologic effects mediated by estrogenic mechanisms. However, clinical and epidemiologic data for infant nutrition sources may provide insights about risks of such substances in human milk and infant formulas.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1241552/

Record 1590

pmcid: PMC1502329

pmid: 16986044

doi:

title: Biochemical (Prostate-Specific Antigen) Relapse: An Oncologist's Perspective.

authors

- Taplin ME

journal: Reviews in urology

journalAbbr: Rev Urol

year: 2003

pubdate: 2003

volume: 5 Suppl 2

issue: Suppl 2

pages: S3-S13

abstract: Consensus has not been reached on the exact definition of biochemical relapse after prostatectomy; individual institution definitions of relapse after prostatectomy range from consecutively rising prostate-specific antigen (PSA) values of > 0.2 to > 0.6 ng/mL. PSA measurements after radiation are even less predictable. PSA level is a sensitive marker of occult prostate-cancer relapse and provides early notification of recurrence, but a PSA relapse does not equal a clinical relapse or death from prostate cancer. Data are reviewed from retrospective, single-institution trials that have clarified features of PSA relapse after both prostatectomy and radiation, such as the PSA doubling time and the time to the first PSA elevation, which are associated with clinical progression. Various options for treatment of biochemical relapse are also reviewed; these include hormone therapy, combined chemohormonal therapy, alternative medicine and dietary tactics, new agents, and future strategies, such as vaccination. Currently, there is no standard treatment for biochemical failure with proven benefit in terms of quality of life, time to metastases, or survival. Current options include observation for patients with long PSA doubling times or comorbid medical issues and standard or nontraditional hormone therapy or a clinical trial for men who desire early therapy or who have rapid PSA doubling times (< 10-12 months). Trials combining the early use of chemotherapy with hormone therapy are promising. Patients should be encouraged to enroll in clinical trials to help establish standards of care.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1502329/

Record 1591

pmcid: PMC1502337

pmid: 16985948

doi:

title: Biochemical (Prostate-Specific Antigen) Relapse: An Oncologist's Perspective.

authors

- Taplin ME

journal: Reviews in urology

journalAbbr: Rev Urol

year: 2003

pubdate: 2003

volume: 5 Suppl 3

issue: Suppl 3

pages: S3-S13

abstract: Consensus has not been reached on the exact definition of biochemical relapse after prostatectomy; individual institution definitions of relapse after prostatectomy range from consecutively rising prostate-specific antigen (PSA) values of > 0.2 to > 0.6 ng/mL. PSA measurements after radiation are even less predictable. PSA level is a sensitive marker of occult prostate-cancer relapse and provides early notification of recurrence, but a PSA relapse does not equal a clinical relapse or death from prostate cancer. Data are reviewed from retrospective, single-institution trials that have clarified features of PSA relapse after both prostatectomy and radiation, such as the PSA doubling time and the time to the first PSA elevation, which are associated with clinical progression. Various options for treatment of biochemical relapse are also reviewed; these include hormone therapy, combined chemohormonal therapy, alternative medicine and dietary tactics, new agents, and future strategies, such as vaccination. Currently, there is no standard treatment for biochemical failure with proven benefit in terms of quality of life, time to metastases, or survival. Current options include observation for patients with long PSA doubling times or comorbid medical issues and standard or nontraditional hormone therapy or a clinical trial for men who desire early therapy or who have rapid PSA doubling times (< 10-12 months). Trials combining the early use of chemotherapy with hormone therapy are promising. Patients should be encouraged to enroll in clinical trials to help establish standards of care.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1502337/

Record 1592

pmcid: PMC1770016

pmid: 12890826

doi: 10.1136/jcp.56.8.639

title: Cirrhosis with steatohepatitis following longterm stilboestrol treatment.

authors

- Cooper L
- Palmer M
- Oien K

journal: Journal of clinical pathology

journalAbbr: J Clin Pathol

year: 2003

pubdate: 2003 Aug

volume: 56

issue: 8

pages: 639

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1770016/

Record 1593

pmcid: PMC1742591

pmid: 12566550

doi: 10.1136/pmj.79.927.36

title: HIV viral suppression in the era of antiretroviral therapy.

authors

- Thaker HK
- Snow MH

journal: Postgraduate medical journal

journalAbbr: Postgrad Med J

year: 2003

pubdate: 2003 Jan

volume: 79

issue: 927

pages: 36-42

abstract: Altogether 42 million people worldwide have been infected with HIV, and 12 million have died over the last 20 years. Effective antiretroviral therapy has lead to sustained HIV viral suppression and immunological recovery in patients who have been infected with the virus. The incidence of AIDS has declined in the Western world with the introduction of effective antiretroviral therapy. Questions on When to start treatment?, What to start with?, How to monitor patients?, remain heavily debated. Adherence to antiretroviral treatment remains the cornerstone of effective treatment, and failure to adhere is the strongest predictor of virological failure. Long term therapy can lead to metabolic complications. Resource poor countries are dealing with difficult issues such as mother to child prevention of HIV transmission. Other treatment options are now available, with the recent introduction of fusion inhibitors, second generation non-nucleoside reverse transcriptase inhibitors, and nucleotide reverse transcriptase inhibitors to clinical practice.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1742591/

Record 1594

pmcid: PMC218761

pmid: 14514888

doi: 10.1073/pnas.2032822100

title: Interpreting hemoglobin and water concentration, oxygen saturation, and scattering measured in vivo by near-infrared breast tomography.

authors

- Srinivasan S
- Pogue BW
- Jiang S
- Dehghani H
- Kogel C
- Soho S
- Gibson JJ
- Tosteson TD

- Poplack SP
- Paulsen KD

journal: Proceedings of the National Academy of Sciences of the United States of America

journalAbbr: Proc Natl Acad Sci U S A

year: 2003

pubdate: 2003 Oct 14

volume: 100

issue: 21

pages: 12349-54

abstract: Near-infrared spectroscopic tomography was used to measure the properties of 24 mammographically normal breasts to quantify whole-breast absorption and scattering spectra and to evaluate which tissue composition characteristics can be determined from these spectra. The absorption spectrum of breast tissue allows quantification of (i) total hemoglobin concentration, (ii) hemoglobin oxygen saturation, and (iii) water concentration, whereas the scattering spectrum provides information about the size and number density of cellular components and structural matrix elements. These property data were tested for correlation to demographic information, including subject age, body mass index, breast size, and radiographic density. Total hemoglobin concentration correlated inversely to body mass index, likely because lower body mass indicates proportionately less fat and more glandular tissue, and glandular tissue contains greater vascularity, hence, more total hemoglobin. Optical scattering was correlated to breast diameter, subject age, and radiographic density. In the radiographic density, fatty breasts had low scattering power and extremely dense breasts had higher values. This observation is consistent with low attenuation of conventional x-rays with fat and higher attenuation in glandular tissues. Optically, fatty tissues have large scatterers leading to a low scattering power, whereas glandular or fibrous tissues have more cellular and collagen-based structures that lead to high scattering power. The study presents correlative data supporting the hypothesis that optical measurements of absorption and scattering can provide physiologically relevant information about breast tissue composition. These breast constituents vary significantly between individuals and can be altered because of changes in breast physiology or pathological state.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC218761/

Record 1595

pmcid: PMC259159

pmid: 14576229

doi: 10.1136/bmj.327.7421.947

title: Safety concerns about breast implants persist.

authors

- Moynihan R

journal: BMJ (Clinical research ed.)

journalAbbr: BMJ

year: 2003

pubdate: 2003 Oct 25

volume: 327

issue: 7421

pages: 947

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC259159/

Record 1596

pmcid: PMC7526745

pmid: 14674745

doi: 10.1021/bi035596s

title: Toxicity of nucleoside analogues used to treat AIDS and the selectivity of the mitochondrial DNA polymerase.

authors

- Lee H
- Hanes J
- Johnson KA

journal: Biochemistry

journalAbbr: Biochemistry

year: 2003

pubdate: 2003 Dec 23

volume: 42

issue: 50

pages: 14711-9

abstract: Incorporation of nucleoside analogues by the mitochondrial DNA polymerase has been implicated as the primary cause underlying many of the toxic side effects of these drugs in HIV therapy. Recent success in reconstituting recombinant human enzyme has afforded a detailed mechanistic analysis of the reactions governing nucleotide selectivity of the polymerase and the proofreading exonuclease. The toxic side effects of nucleoside analogues are correlated with the kinetics of incorporation by the mitochondrial DNA polymerase, varying over 6 orders of magnitude in the sequence zalcitabine (ddC) > didanosine (ddI metabolized to ddA) > stavudine (d4T) >> lamivudine (3TC) > tenofovir (PMPA) > zidovudine (AZT) > abacavir (metabolized to carbovir, CBV). In this review, we summarize our current efforts to examine the mechanistic basis for nucleotide selectivity by the mitochondrial DNA polymerase and its role in mitochondrial toxicity of nucleoside analogues used to treat AIDS and other viral infections. We will also discuss the promise and underlying challenges for the development of new analogues with lower toxicity.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7526745/

Record 1597

pmcid: PMC137592

pmid: 12383352

doi: 10.1186/1471-2407-2-26

title: Male gynecomastia and risk for malignant tumours--a cohort study.

authors

- Olsson H
- Bladstrom A
- Alm P

journal: BMC cancer

journalAbbr: BMC Cancer

year: 2002

pubdate: 2002 Oct 16

volume: 2

issue:

pages: 26

abstract: BACKGROUND: Men with gynecomastia may suffer from absolute or relative estrogen excess and their risk of different malignancies may be increased. We tested whether men with gynecomastia were at greater risk of developing cancer. METHODS: A cohort was formed of all the men having a histopathological diagnosis of gynecomastia at the Department of Pathology, University of Lund, following an operation for either uni- or bilateral breast enlargement between 1970-1979. All possible causes of gynecomastia were accepted, such as endogenous or exogenous hormonal exposure as well as cases of unknown etiology. Prior to diagnosis of gynecomastia eight men had a diagnosis of prostate carcinoma, two men a diagnosis of unilateral breast cancer and one had Hodgkin's disease. These patients were included in the analyses. The final cohort of 446 men was matched to the Swedish Cancer Registry, Death Registry and General Population Registry. RESULTS: At the end of the follow up in December 1999, the cohort constituted 8375.2 person years of follow-up time. A total of 68 malignancies versus 66.07 expected were observed; SIR = 1.03 (95% CI 0.80-1.30). A significantly increased risk for testicular cancer; SIR = 5.82 (95% CI 1.20-17.00) and squamous cell carcinoma of the skin; SIR = 3.21 (95% CI 1.71-5.48) were noted. The increased risk appeared after 2 years of follow-up. A non-significantly increased risk for esophageal cancer was also seen while no new cases of male breast cancer were observed. However, in the prospective cohort, diagnostic operations for gynecomastia may substantially have reduced this risk CONCLUSIONS: There is a significant increased risk of testicular cancer and squamous cell carcinoma of the skin in men who have been operated on for gynecomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC137592/

Record 1598

pmcid: PMC4925338

pmid: 27407396

doi: 10.1016/S0377-1237(02)80145-9

title: Reduction Mammaplasty Using McKissock Vertical Bipedicle Technique.

authors

- Bhatnagar V

journal: Medical journal, Armed Forces India

journalAbbr: Med J Armed Forces India

year: 2002

pubdate: 2002 Jul

volume: 58

issue: 3

pages: 259-61

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC4925338/

Record 1599

pmcid: PMC1281630

pmid: 11461980

doi: 10.1177/014107680109400803

title: Aldosterone as a determinant of cardiovascular and renal dysfunction.

authors

- Epstein M

journal: Journal of the Royal Society of Medicine

journalAbbr: J R Soc Med

year: 2001

pubdate: 2001 Aug

volume: 94

issue: 8

pages: 378-83

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1281630/

Record 1600

pmcid: PMC1572718

pmid: 11309248

doi: 10.1038/sj.bjp.0703974

title: Direct inhibitory effect of digitalis on progesterone release from rat granulosa cells.

authors

- Chen JJ
- Wang PS
- Chien EJ
- Wang SW

journal: British journal of pharmacology

journalAbbr: Br J Pharmacol

year: 2001

pubdate: 2001 Apr

volume: 132

issue: 8

pages: 1761-8

abstract: Digoxin (10(-7) - 10(-5) M) or digitoxin (10(-7) - 10(-5) M) decreased the basal and human chorionic gonadotropin (hCG)-stimulated release of progesterone from rat granulosa cells. Digoxin (10(-5) M) or digitoxin (10(-5) M) attenuated the stimulatory effects of forskolin and 8-bromo-cyclic 3' : 5'-adenosine monophosphate (8-Br-cAMP) on progesterone release from rat granulosa cells. Digoxin (10(-5) M) or digitoxin (10(-5) M) inhibited cytochrome P450 side chain cleavage enzyme (cytochrome P450(scc)) activity (conversion of 25-hydroxyl cholesterol to pregnenolone) in rat granulosa cells but did not influence the activity of 3beta-hydroxysteroid dehydrogenase (3beta-HSD). Neither progesterone production nor P450scc activity in rat granulosa cells was altered by the administration of ouabain. Digoxin (10(-5) M) or digitoxin (10(-5) M), but not ouabain, decreased the expression of P450scc and steroidogenic acute regulatory (StAR) protein in rat granulosa cells. The present results suggest that digoxin and digitoxin decrease the progesterone release by granulosa cells via a Na(+),K(+)-ATPase-independent mechanism involving the inhibition of post-cyclic AMP pathway, cytochrome P450scc and StAR protein functions.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1572718/

Record 1601

pmcid: PMC7094406

pmid: 11179239

doi: 10.1053/gast.2001.22427

title: Emerging concepts in gastrointestinal aspects of HIV-1 pathogenesis and management.

authors

- Janoff EN
- Smith PD

journal: Gastroenterology

journalAbbr: Gastroenterology

year: 2001

pubdate: 2001 Feb

volume: 120

issue: 3

pages: 607-21

abstract: GASTROENTROLOGY 2001;120:607-621

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC7094406/

Record 1602

pmcid: PMC1744352

pmid: 11463934

doi: 10.1136/sti.77.4.293

title: Female low sexual desire and sexually transmitted infections.

authors

- Goldmeier D

journal: Sexually transmitted infections

journalAbbr: Sex Transm Infect

year: 2001

pubdate: 2001 Aug

volume: 77

issue: 4

pages: 293-4

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1744352/

Record 1603

pmcid: PMC1407752

pmid: 11291533

doi:

title: Gynecomastia induced by treatment with tandospirone, a 5-HT1A agonist.

authors

- Kaneda Y

- Morimoto T
- Fujii A

journal: Journal of psychiatry & neuroscience : JPN

journalAbbr: J Psychiatry Neurosci

year: 2001

pubdate: 2001 Mar

volume: 26

issue: 2

pages: 152-3

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1407752/

Record 1604

pmcid: PMC1173269

pmid:

doi:

title: Health department misled women over breast implant risks, groups claim.

authors

- Kmietowicz Z

journal: BMJ : British Medical Journal

journalAbbr: BMJ

year: 2001

pubdate: 2001 Mar 31

volume: 322

issue: 7289

pages: 756

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1173269/

Record 1605

pmcid: PMC1119343

pmid:

doi:

title: Making and breaking news.

authors

journal: BMJ : British Medical Journal

journalAbbr: BMJ

year: 2001

pubdate: 2001 Jan 13

volume: 322

issue: 7278

pages: 0

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1119343/

Record 1606

pmcid: PMC1119342

pmid:

doi:

title: Making and breaking news.

authors

journal: BMJ : British Medical Journal

journalAbbr: BMJ

year: 2001

pubdate: 2001 Jan 13

volume: 322

issue: 7278

pages: 0

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1119342/

Record 1607

pmcid: PMC1120371

pmid: 11375217

doi: 10.1136/bmj.322.7297.1263

title: The BMJ 's ethics committee is open for business.

authors

- Tonks A
- Smith AM
- Smith R

journal: BMJ (Clinical research ed.)

journalAbbr: BMJ

year: 2001

pubdate: 2001 May 26

volume: 322

issue: 7297

pages: 1263-4

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1120371/

Record 1608

pmcid: PMC1741519

pmid: 10684334

doi: 10.1136/pmj.76.893.175

title: A case of intestinal obstruction.

authors

- Michell NP
- Routledge H
- Chesner IM

journal: Postgraduate medical journal

journalAbbr: Postgrad Med J

year: 2000

pubdate: 2000 Mar

volume: 76

issue: 893

pages: 175-6

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1741519/

Record 1609

pmcid: PMC1741527

pmid: 10684335

doi: 10.1136/pmj.76.893.177

title: Isosexual precocity: uncommon presentation of a common disorder.

authors

- Bhansali A
- Kashyap A
- Lodha S
- Kotwal N
- Ganapathi B
- Mittal BR
- Dash RJ

journal: Postgraduate medical journal

journalAbbr: Postgrad Med J

year: 2000

pubdate: 2000 Mar

volume: 76

issue: 893

pages: 177-8

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1741527/

Record 1610

pmcid: PMC1070801

pmid: 10734809

doi: 10.1136/ewjm.172.3.182

title: Myths and variations in normal pubertal development.

authors

- Nakamoto JM

journal: The Western journal of medicine

journalAbbr: West J Med

year: 2000

pubdate: 2000 Mar

volume: 172

issue: 3

pages: 182-5

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1070801/

Record 1611

pmcid: PMC1231219

pmid: 10738450

doi:

title: Neonatal hypernatremic dehydration associated with breast-feeding malnutrition: a retrospective survey.

authors

- Livingstone VH
- Willis CE
- Abdel-Wareth LO
- Thiessen P
- Lockitch G

journal: CMAJ : Canadian Medical Association journal = journal de l'Association medicale canadienne

journalAbbr: CMAJ

year: 2000

pubdate: 2000 Mar 7

volume: 162

issue: 5

pages: 647-52

abstract: BACKGROUND: Hypernatremic dehydration in neonates is a potentially devastating condition. Recent reports have identified breast-feeding malnutrition as a key factor in its pathophysiology. METHODS: Using a theoretical framework for breast-feeding kinetics, a retrospective chart review of all neonates less than 28 days of age who were seen at either British Columbia's Children's Hospital or the Vancouver Breastfeeding Centre between 1991-1994 was conducted to identify and classify possible causes of breast-feeding malnutrition among neonates who developed hypernatremic dehydration. RESULTS: Twenty-one cases hypernatremic

dehydration were identified. Infant weight loss ranged from 8% to 30% of birth weight, and serum sodium levels ranged from 146 mmol/L to 207 mmol/L. In each case, maternal or infant factors (e.g., poor breast-feeding technique, lactation failure following postpartum hemorrhage and infant suckling disorders associated with cleft palate or ankyloglossia) that could interfere with either lactation or breast-feeding dynamics and account for insufficient breast milk intake were identified. INTERPRETATION: Prenatal and in-hospital screening for maternal and infant risk factors for breast-feeding malnutrition combined with early postpartum follow-up to detect excessive infant weight loss are important for the prevention of neonatal hypernatremic dehydration.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1231219/

Record 1612

pmcid: PMC13613310

pmid: 10796730

doi: 10.1002/14651858.CD001018

title: Oestrogen and progestogen hormone replacement therapy for peri-menopausal and post-menopausal women: weight and body fat distribution.

authors

- Norman RJ
- Flight IH
- Rees MC

journal: The Cochrane database of systematic reviews

journalAbbr: Cochrane Database Syst Rev

year: 2000

pubdate: 2000

volume: 1999

issue: 2

pages: CD001018

abstract: BACKGROUND: Hormone replacement therapy (HRT) is commonly prescribed to treat menopausal symptoms and to prevent post-menopausal bone loss. However, many women are concerned about hormonal replacement therapy because they believe that such treatment will result in weight gain. The effect of HRT on weight and body fat distribution has not yet been examined in systematic reviews. It is an important topic since many women decline oestrogen therapy due to their concerns about resultant weight gain, and thus forego its potential therapeutic benefits. OBJECTIVES: To evaluate the effect of unopposed oestrogen or combined oestrogen and progestogen hormone replacement therapy (HRT) upon the weight and body fat distribution of perimenopausal and postmenopausal women. SEARCH STRATEGY: The search strategy of the Menstrual Disorders and Subfertility Group was used for the identification of randomised controlled trials (RCTs). Computerised searches of MEDLINE, EMBASE, Current Contents, Biological Abstracts and CINAHL were performed. Attempts were made to identify trials from citation lists of review articles and relevant papers already obtained. In most cases, first authors of each eligible trial were contacted for additional information. All those trials that had been located as at August 1998 were examined for eligibility. SELECTION CRITERIA: All randomised, placebo or no treatment controlled trials that detailed the effect of HRT on weight or body fat distribution, including studies where HRT was combined with other therapy such as diet, supplements or exercise. Studies were eligible for consideration even though the main focus of the trial may have been on another aspect of HRT. Previous HRT use should have ceased at least one month (in the case of patches, cream or gel) or three months (for oral preparations or subcutaneous pellets) before commencement of the study. DATA COLLECTION AND ANALYSIS: Twenty two RCTs were identified that fulfilled the inclusion criteria for this review. The results of one trial were not available in a form that allowed it to be included in the meta-analysis; however, it has been included in the text of the review for discussion. Twenty four RCTs are awaiting assessment pending additional information from first authors. Two reviewers extracted the data independently, and the weighted mean differences for continuous outcomes were estimated from the data. Results for

unopposed oestrogen and combined oestrogen were analysed separately, and the effect of each treatment regimen on body weight, BMI, waist-hip ratio, fat mass and skinfold measurement was examined where available. The effect of differing dosage levels on these parameters was also examined. MAIN RESULTS: Outcomes were evaluated separately for unopposed oestrogen and oestrogen/progestogen regimens. Statistical analysis was performed using the weighted mean difference for continuous outcomes as recommended by the Cochrane Menstrual Disorders and Subfertility Group. No statistically significant difference was found in mean weight gain between those using unopposed oestrogen and non-HRT users (0.66 kg, 95% CI -0.62, 1.93). No significant difference was found in mean weight gain between those using oestrogen/progestogen therapy and non-HRT users (-0.47 kg, 95% CI -1.63, 0.69). Insufficient data exist to enable meta-analysis of the effect of unopposed oestrogen on BMI. The reviewers found no statistically significant difference in mean BMI increase between those using oestrogen/progestogen and non-HRT users (-0.50, 95% CI -1.06, 0.06). Insufficient data exist to enable meta-analysis of the effect of HRT on waist-hip ratio, fat mass or skinfold thickness. REVIEWER'S CONCLUSIONS: There is evidence of no effect of unopposed oestrogen or combined oestrogen on body weight, indicating that these regimens do not cause extra weight gain in addition to that normally gained at menopause. (ABSTRACT TRUNCATED)

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC13613310/

Record 1613

pmcid: PMC1495721

pmid: 10808380

doi: 10.1007/s11606-000-0019-1

title: Society of General Internal Medicine 23rd annual meeting. Boston, Massachusetts, USA. May 4-6, 2000. Abstracts.

authors

journal: Journal of general internal medicine

journalAbbr: J Gen Intern Med

year: 2000

pubdate: 2000 Apr

volume: 15 Suppl 1

issue: Suppl 1

pages: 23-242

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1495721/

Record 1614

pmcid: PMC3173037

pmid: 10683055

doi: 10.1001/jama.283.6.763

title: Testosterone replacement and resistance exercise in HIV-infected men with weight loss and low testosterone levels.

authors

- Bhasin S
- Storer TW
- Javanbakht M
- Berman N

- Yarasheski KE
- Phillips J
- Dike M
- Sinha-Hikim I
- Shen R
- Hays RD
- Beall G

journal: JAMA

journalAbbr: JAMA

year: 2000

pubdate: 2000 Feb 9

volume: 283

issue: 6

pages: 763-70

abstract: CONTEXT: Previous studies of testosterone supplementation in HIV-infected men failed to demonstrate improvement in muscle strength. The effects of resistance exercise combined with testosterone supplementation in HIV-infected men are unknown. OBJECTIVE: To determine the effects of testosterone replacement with and without resistance exercise on muscle strength and body composition in HIV-infected men with low testosterone levels and weight loss. DESIGN AND SETTING: Placebo-controlled, double-blind, randomized clinical trial conducted from September 1995 to July 1998 at a general clinical research center. PARTICIPANTS: Sixty-one HIV-infected men aged 18 to 50 years with serum testosterone levels of less than 12.1 nmol/L (349 ng/dL) and weight loss of 5% or more in the previous 6 months, 49 of whom completed the study. INTERVENTIONS: Participants were randomly assigned to 1 of 4 groups: placebo, no exercise (n = 14); testosterone enanthate (100 mg/wk intramuscularly), no exercise (n = 17); placebo and exercise (n = 15); or testosterone and exercise (n = 15). Treatment duration was 16 weeks. MAIN OUTCOME MEASURES: Changes in muscle strength, body weight, thigh muscle volume, and lean body mass compared among the 4 treatment groups. RESULTS: Body weight increased significantly by 2.6 kg (P<.001) in men receiving testosterone alone and by 2.2 kg (P = .02) in men who exercised alone but did not change in men receiving placebo alone (-0.5 kg; P = .55) or testosterone and exercise (0.7 kg; P = .08). Men treated with testosterone alone, exercise alone, or both experienced significant increases in maximum voluntary muscle strength in leg press (range, 22%-30%), leg curls (range, 18%-36%), bench press (range, 19%-33%), and latissimus pulls (range, 17%-33%). Gains in strength in all exercise categories were greater in men assigned to the testosterone-exercise group or to the exercise-alone group than in those assigned to the placebo-alone group. There was a greater increase in thigh muscle volume in men receiving testosterone alone (mean change, 40 cm3; P<.001 vs zero change) or exercise alone (62 cm3; P = .003) than in men receiving placebo alone (5 cm3; P = .70). Average lean body mass increased by 2.3 kg (P = .004) and 2.6 kg (P<.001), respectively, in men who received testosterone alone or testosterone and exercise but did not change in men receiving placebo alone (0.9 kg; P = .21). Hemoglobin levels increased in men receiving testosterone but not in those receiving placebo. CONCLUSION: Our data suggest that testosterone and resistance exercise promote gains in body weight, muscle mass, muscle strength, and lean body mass in HIV-infected men with weight loss and low testosterone levels. Testosterone and exercise together did not produce greater gains than either intervention alone.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3173037/

Record 1615

pmcid: PMC1758191

pmid: 10448360

doi: 10.1136/sti.75.2.90

title: British HIV Association guidelines for prescribing antiretroviral therapy in pregnancy (1998).

authors

- Taylor GP
- Lyall EG
- Mercey D
- Smith R
- Chester T
- Newell ML
- Tudor-Williams G

journal: Sexually transmitted infections

journalAbbr: Sex Transm Infect

year: 1999

pubdate: 1999 Apr

volume: 75

issue: 2

pages: 90-7

abstract: The aim of antiretroviral therapy in pregnancy is to deliver a healthy uninfected child to a healthy mother, without prejudicing the future treatment opportunities of the mother. The use of zidovudine monotherapy rapidly became standard practice once it had been shown to reduce by 67% mother to child transmission in women with CD4+ lymphocyte counts above 200 x 10(6)/l. High rates of transmission are seen when maternal disease is advanced (high viral load, low CD4+ lymphocyte counts) despite zidovudine. In these women highly active antiretroviral therapy gives the best prospect for prolonged health and it is anticipated that reducing plasma viral load below the limits of detection will further reduce transmission rates. However, safety data for antiretroviral therapy in pregnancy are limited and each additional treatment exposes a significant proportion of uninfected infants to potential long term hazards. Where maternal therapy is not indicated and the sole objective of treatment is to reduce mother to child transmission, recent data suggest that short course zidovudine (especially in conjunction with prelabour caesarean section) is a reasonable option. This may minimise the emergence of viruses with reduced sensitivity to zidovudine and preserve maternal options for later therapy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1758191/

Record 1616

pmcid: PMC3456696

pmid: 10609596

doi: 10.1007/BF02351504

title: Evaluating supervised HAART in late-stage HIV among drug users: a preliminary report.

authors

- Greenberg B
- Berkman A
- Thomas R
- Hoos D
- Finkelstein R
- Astemborski J
- Vlahov D

journal: Journal of urban health : bulletin of the New York Academy of Medicine

journalAbbr: J Urban Health

year: 1999

pubdate: 1999 Dec

volume: 76

issue: 4

pages: 468-80

abstract: OBJECTIVE: To examine response to highly active antiretroviral therapy (HAART) among a sample of treatment-experienced patients in the late stage of human immunodeficiency virus (HIV) infection in residential health care facilities (RHCFs) in New York City facilities designated for HIV/AIDS (acquired immunodeficiency syndrome) when access and adherence are maximized. METHODS: Medical record review of 111 patients. RESULTS: Demographics were mean age 42 years; 58% male; 60% African-American; 31% Hispanic; 57% injection drug users (IDUs); 23% with history of dementia; 52% hepatitis C virus (HCV) antibody seropositive; 80% on HAART, of whom 18% had lipodystrophy. Of 88 patients on HAART, 52% had a decreased viral load (>1/2 log) versus 13% of 23 not on HAART (P<.05); a >1/2 log viral load increase was seen in 8% and 35%, respectively (P<.05). Those with viral load increase were more likely than those with stable/decreased viral load to be IDUs (71% vs. 64%) and to have HCV seropositivity (86% vs. 53%), even with similar initial CD4+ cell count, viral load, and follow-up time. CONCLUSION: In a predominantly minority IDU population who are treatment experienced, 50% of the patients successfully responded to treatment with supervised therapy. The RHCFs in New York City provide a unique opportunity to examine further factors associated with response to HAART in an environment in which medication administration and adherence are maximized and monitored carefully.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3456696/

Record 1617

pmcid: PMC2328407

pmid: 10099811

doi:

title: Hope for HIV patients. News from the 12th International Conference on HIV and AIDS.

authors

- Elliott EB

journal: Canadian family physician Medecin de famille canadien

journalAbbr: Can Fam Physician

year: 1999

pubdate: 1999 Mar

volume: 45

issue:

pages: 829-31

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2328407/

Record 1618

pmcid: PMC1477529

pmid: 16985794

doi:

title: New Strategies for Bladder and Prostate Cancer, BPH Explored by South Central Urologists: Highlights from the South Central Section of the American Urological Association January 16-20, 1999, Cancun, Mexico.

authors

- Roehrborn CG

journal: Reviews in urology

journalAbbr: Rev Urol

year: 1999

pubdate: 1999 Fall

volume: 1

issue: 4

pages: 193-9

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1477529/

Record 1619

pmcid: PMC6761906

pmid: 15706796

doi:

title: Alcohol's effects on male reproduction.

authors

- Emanuele MA
- Emanuele NV

journal: Alcohol health and research world

journalAbbr: Alcohol Health Res World

year: 1998

pubdate: 1998

volume: 22

issue: 3

pages: 195-201

abstract: The male reproductive system consists of the hypothalamus, the anterior pituitary gland, and the testes. Alcohol can interfere with the function of each of these components, thereby causing impotence, infertility, and reduced male secondary sexual characteristics. In the testes, alcohol can adversely affect the Leydig cells, which produce and secrete the hormone testosterone. Studies found that heavy alcohol consumption results in reduced testosterone levels in the blood. Alcohol also impairs the function of the testicular Sertoli cells that play an important role in sperm maturation. In the pituitary gland, alcohol can decrease the production, release, and/or activity of two hormones with critical reproductive functions, luteinizing hormone and follicle-stimulating hormone. Finally, alcohol can interfere with hormone production in the hypothalamus.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC6761906/

Record 1620

pmcid: PMC2278276

pmid: 9640524

doi:

title: Alternative treatments for menopausal symptoms. Systematic review of scientific and lay literature.

authors

- Seidl MM
- Stewart DE

journal: Canadian family physician Medecin de famille canadien

journalAbbr: Can Fam Physician

year: 1998

pubdate: 1998 Jun

volume: 44

issue:

pages: 1299-308

abstract: OBJECTIVE: To review the scientific literature on common alternative remedies for treatment of symptoms attributed to menopause and to contrast this with available lay literature. QUALITY OF EVIDENCE: Scientific articles were identified by searching MEDLINE, CINAHL, and HEALTH databases from 1966 to mid-1997 for English-language articles. More than 200 references were reviewed; 85 were selected for citation based on specific reference to alternative medicine for symptoms commonly attributed to menopause (e.g., hot flashes), to the effects of changing estrogen levels (e.g., irregular menses, vaginal dryness), and to reported side effects of the treatments. MAIN FINDINGS: The scientific literature was categorized under the headings nutritional supplements, herbal remedies, homeopathic remedies, and physical approaches. Some scientific evidence of the safety and efficacy of alternative treatments during menopause was uncovered, with the strongest evidence emerging in favour of phytoestrogens, which occur in high concentrations as isoflavones in soy products. CONCLUSIONS: In available controlled studies, the strongest data support phytoestrogens for their role in diminishing menopausal symptoms related to estrogen deficiency and for possible protective effects on bones and the cardiovascular system. Randomized controlled trials, standardization of dosage, and accurate safety and efficacy labeling are required to ensure proper use of alternative remedies.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2278276/

Record 1621

pmcid: PMC1752512

pmid: 10070277

doi: 10.1136/ard.57.12.748

title: Chondrocalcinosis and Gitelman's syndrome. A new association?

authors

- Cobeta-Garcia JC
- Gascón A
- Iglesias E
- Estopiñán V

journal: Annals of the rheumatic diseases

journalAbbr: Ann Rheum Dis

year: 1998

pubdate: 1998 Dec

volume: 57

issue: 12

pages: 748-9

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1752512/

Record 1622

pmcid: PMC1296879

pmid: 9849522

doi: 10.1177/014107689809100910

title: Nipple discharge in a teenager.

authors

- Mudan SS
- Ibrahim AE
- Wise M
- Perry PM

journal: Journal of the Royal Society of Medicine

journalAbbr: J R Soc Med

year: 1998

pubdate: 1998 Sep

volume: 91

issue: 9

pages: 490-1

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1296879/

Record 1623

pmcid: PMC1752517

pmid: 10070278

doi: 10.1136/ard.57.12.749

title: Parotid lipomatosis in HIV positive patients: a new clinical disorder associated with protease inhibitors.

authors

- Olivé A
- Salavert A
- Manríquez M
- Clotet B
- Moragas A

journal: Annals of the rheumatic diseases

journalAbbr: Ann Rheum Dis

year: 1998

pubdate: 1998 Dec

volume: 57

issue: 12

pages: 749

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1752517/

Record 1624

pmcid: PMC1051464

pmid: 9783712

doi: 10.1136/jmg.35.10.852

title: Seminoma in a postmenopausal woman with a Y;15 translocation in peripheral blood lymphocytes and a t(Y;15)/45,X Turner mosaic pattern in skin fibroblasts.

authors

- Hoshi N
- Fujita M
- Mikuni M
- Fujino T
- Okuyama K
- Handa Y
- Yamada H
- Sagawa T
- Hareyama H
- Nakahori Y
- Fujieda K
- Kant JA
- Nagashima K
- Fujimoto S

journal: Journal of medical genetics

journalAbbr: J Med Genet

year: 1998

pubdate: 1998 Oct

volume: 35

issue: 10

pages: 852-6

abstract: We report an unusual case of a 55 year old Japanese woman with a seminoma but relatively normal menses. The patient was a phenotypic female with late onset menarche (18 years of age), who was amenorrhoeic for the first year, followed by menses of one to three days' slight flow with dysmenorrhoea, but an otherwise normal menstrual history. A typical seminoma was removed from the left adnexal region and an immature testis was identified separately as an associated right adnexal mass. Repeated karyotypic studies on peripheral blood lymphocyte cultures showed only 46,X,-Y,t(Y;15)(q12;p13). Cytogenetic examination of the patient's younger brother, who had fathered three healthy children, showed an identical karyotype. Mosaicism of 46,X,-Y,t(Y;15)(q12;p13)/45,X cell lines was found in skin samples from the patient's elbow and genital regions, although there were no clinical stigmata of Turner syndrome. An androgen receptor binding assay of cultured genital skin fibroblasts was negative. Molecular analysis using Southern blot hybridisation, PCR, and direct DNA sequencing showed that neither the patient nor her brother had a detectable deletion or other abnormalities of Y chromosome sequences, including the SRY (sex determining region of the Y chromosome) gene sequence. These findings suggest that Turner mosaicism of the 45,X cell line may have contributed to this atypical presentation in an XY female, although we cannot exclude abnormalities of other genes related to sex differentiation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1051464/

Record 1625

pmcid: PMC1308438

pmid: 9847926

doi:

title: Silicone gel breast implant adverse event reports to the Food and Drug Administration, 1984-1995.

authors

- Brown SL
- Parmentier CM
- Woo EK
- Vishnuvajjala RL
- Headrick ML

journal: Public health reports (Washington, D.C. : 1974)

journalAbbr: Public Health Rep

year: 1998

pubdate: 1998 Nov-Dec

volume: 113

issue: 6

pages: 535-43

abstract: OBJECTIVES: To characterize the adverse event reports on silicone gel breast implants (SGBIs), including death reports, submitted to the Food and Drug Administration (FDA) from 1984 through 1995 and to analyze changes in the type and complexity of reports following extensive media coverage of breast implants. METHODS: The authors analyzed mandatory and voluntary reports from the adverse events reporting system for medical devices at the FDA. RESULTS: In 1988, adverse event reports related to SGBIs accounted for 2.4% of the 14,473 mandatory reports entered into the FDA database on medical devices. In 1992, SGBI-related reports accounted for 30.3% of the total 66,476 mandatory reports of adverse events. The most frequently reported adverse event in 1988, before the widespread publicity on breast implants, was implant burst or rupture. In contrast, in 1992 the most frequently reported event was reaction, a term used to describe a range of adverse effects. CONCLUSIONS: The numbers of mandatory and voluntary reports of SGBI-related adverse events increased exponentially, as did the complexity of the reports, following publicity over the lack of safety data on breast implants and a short voluntary moratorium on their sale. A significant proportion of reports lacked information on specific medical symptoms or diagnoses.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1308438/

Record 1626

pmcid: PMC1752514

pmid: 10070279

doi: 10.1136/ard.57.12.749a

title: Ultrasound guided injection of plantar fasciitis.

authors

- Quinn M
- Gough A

journal: Annals of the rheumatic diseases

journalAbbr: Ann Rheum Dis

year: 1998

pubdate: 1998 Dec

volume: 57

issue: 12

pages: 749-50

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1752514/

Record 1627

pmcid: PMC2223802

pmid: 9218739

doi: 10.1038/bjc.1997.342

title: Epirubicin combined with estramustine phosphate in hormone-resistant prostate cancer: a phase II study.

authors

- Hernes EH
- Fosså SD
- Vaage S
- Ogreid P
- Heilo A
- Paus E

journal: British journal of cancer

journalAbbr: Br J Cancer

year: 1997

pubdate: 1997

volume: 76

issue: 1

pages: 93-9

abstract: Twenty-four assessable patients with hormone-resistant prostate cancer (HRPC) were to receive daily doses of oral estramustine phosphate (EMP), 10 mg kg(-1), and intravenous epirubicin (EPR) infusions, 100 mg m(-2), every third week up to a cumulative dose of 500 mg m(-2). Biochemical response [> or = 50% reduction in pretreatment serum prostate-specific antigen (PSA) after three cycles of > or = 3 weeks' duration] was demonstrated in 13 of 24 patients included (54%). No objective response (WHO criteria) was observed, although seven of nine evaluable patients achieved a > or = 50% serum PSA reduction. Subjective improvement (pain score, performance status) occurred in 7 of 24 patients, whereas nine patients progressed subjectively. There was no correlation between subjective and biochemical response. Biochemical progression (> or = 50% increase of nadir PSA) occurred after a median of 12 weeks. All but two patients were alive after a median follow-up time of 8.7 months for surviving patients (range 3.3-13.2). Eight patients experienced grade 3/4 leucopenia, with no indication of cumulative myelosuppression. Cardiovascular toxicity was experienced by four patients. Two patients developed angioedema twice, in one patient requiring hospitalization at the intensive ward. Based on this limited series, the combination of EPR and EMP in patients with HRPC is tolerable and appears to be effective in terms of significant PSA reduction. The results warrant further investigations of the two drugs and, in particular, of the clinical significance of > or = 50% PSA decrease in patients with HRPC.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2223802/

Record 1628

pmcid: PMC2431416

pmid: 9338032

doi: 10.1136/pgmj.73.861.428

title: Male breast tuberculosis.

authors

- Jaideep C
- Kumar M
- Khanna AK

journal: Postgraduate medical journal

journalAbbr: Postgrad Med J

year: 1997

pubdate: 1997 Jul

volume: 73

issue: 861

pages: 428-9

abstract: Tuberculosis of the breast is rare and tuberculosis of the male breast is not a recognised entity. We describe a man with tuberculosis of the breast which was clinically thought to be a malignancy.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2431416/

Record 1629

pmcid: PMC3950144

pmid: 8697326

doi:

title: Bilateral primary breast lymphoma in pregnancy: a case report and literature review.

authors

- Kirkpatrick AW
- Bailey DJ
- Weizel HA

journal: Canadian journal of surgery. Journal canadien de chirurgie

journalAbbr: Can J Surg

year: 1996

pubdate: 1996 Aug

volume: 39

issue: 4

pages: 333-5

abstract: Primary lymphoma of the breast is an uncommon malignant breast tumour, which is seldom distinguished preoperatively from other more common forms of breast cancer. The management differs from the more typical adenocarcinoma of breast in that the emphasis is on systemic chemotherapy. A young white woman presented during the 3rd trimester with bilateral breast enlargement, which failed to involute post partum. Subsequent biopsy revealed primary breast lymphoma of the large cell immunoblastic type. She

received systemic chemotherapy and was alive and well with no evidence of recurrence 28 months later.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3950144/

Record 1630

pmcid: PMC2607994

pmid: 8764526

doi:

title: Cancer of the male breast: a review.

authors

- Williams WL Jr
- Powers M
- Wagman LD

journal: Journal of the National Medical Association

journalAbbr: J Natl Med Assoc

year: 1996

pubdate: 1996 Jul

volume: 88

issue: 7

pages: 439-43

abstract: A review of 20 cases of male breast cancer in 17 patients from 1959 to 1990 was performed. The median age at presentation was 53.3 years (range: 29 to 79). At the time of diagnosis, 30% (6) patients were stage I, 65% (13) stage II, and 5% (1) stage IV. Surgery was the initial form of therapy in all cases either as a radical mastectomy, modified radical mastectomy, or total mastectomy. The median disease-free survival was 4.8 years with a 5-year disease-free survival of 41%, and with a 5-year overall survival of 47%. Seven patients are alive with no evidence of disease, two are alive with disease, five have died of disease, and two died of other causes. Estrogen receptor (ER) and progesterone receptor (PR) assays were performed on the tumors of 10 patients, with 80% being ER positive and 70% PR positive. The median disease-free survival for ER positive patients was 6 months with a 5-year disease-free survival of 12.5%. The overall median survival for this group was 2.9 years with an overall 5-year survival of 25%. In this review there was a high percentage of patients who were ER positive. The positive receptor status had value in predicting decreased survival.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2607994/

Record 1631

pmcid: PMC2146478

pmid: 8704491

doi:

title: Chest pain in children.

authors

- Leung AK
- Robson WL
- Cho H

journal: Canadian family physician Medecin de famille canadien

journalAbbr: Can Fam Physician

year: 1996

pubdate: 1996 Jun

volume: 42

issue:

pages: 1156-60, 1163-4

abstract: Chest pain is usually a benign symptom in children. The most common identifiable causes are musculoskeletal. Often, no cause can be identified. Cardiac disorders are uncommon causes of chest pain children. Most causes can be diagnosed from history and physical examination. Treatment should be directed at the underlying cause. For idiopathic chest pain, reassurance and regular follow-up examinations are important.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2146478/

Record 1632

pmcid: PMC2970881

pmid: 21584141

doi:

title: Delusions of pregnancy - a report of four cases.

authors

- Dutta S
- Vankar GK

journal: Indian journal of psychiatry

journalAbbr: Indian J Psychiatry

year: 1996

pubdate: 1996 Oct

volume: 38

issue: 4

pages: 254-6

abstract: We report four cases (three women, one man) having delusions of pregnancy seen over a four month period at a short - stay in-patient facility in a General Hospital Psychiatry Department. None had organic factors and all could be explained on the basis of psychological mechanisms. Delusions of pregnancy may not be as uncommon as was previously believed.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2970881/

Record 1633

pmcid: PMC2549093

pmid: 7711531

doi: 10.1136/bmj.310.6981.686

title: Rationing: the debate we have to have.

authors

- Smith R

journal: BMJ (Clinical research ed.)

journalAbbr: BMJ

year: 1995

pubdate: 1995 Mar 18

volume: 310

issue: 6981

pages: 686

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2549093/

Record 1634

pmcid: PMC1128201

pmid: 7795738

doi: 10.1136/oem.52.4.233

title: Study of morbidity of personnel with potential exposure to vinclozolin.

authors

- Zober A
- Hoffmann G
- Ott MG
- Will W
- Germann C
- van Ravenzwaay B

journal: Occupational and environmental medicine

journalAbbr: Occup Environ Med

year: 1995

pubdate: 1995 Apr

volume: 52

issue: 4

pages: 233-41

abstract: OBJECTIVES: To examine internal exposure and targeted health outcomes of employees exposed to 3-(3,5-dichlorophenyl)-5-methyl-5-vinyl-1,3-oxazolidine-2,4-dione; chemical abstracts service (CAS) number: 50471-44-8 (vinclozolin). METHODS: A cross sectional study of 67 men exposed to vinclozolin for one to 13 years during synthesis and formulation operations and 52 controls. Biomonitoring was based on determination of urinary metabolites that contained a 3,5-dichloroaniline (3,5-DCA) moiety. Targeted health endpoints were the same as in previous subchronic and chronic animal studies--namely, reversible changes in the concentrations of hormones of the adrenocorticotrophic and gonadotrophic feedback systems, signs of liver injury, haemolytic anaemia, cataract formation (uniquely in rats), and hormonally induced hyperplasia and tumours at high doses. The clinical investigation consisted of a medical and occupational history questionnaire, physical examination, laboratory determinations (including testosterone, LH, and FSH measurements), ultrasonography of the liver and prostate, a detailed eye examination, and routine spirometry. RESULTS: The mean 3,5-DCA concentration for two thirds of the study group exceeded an equivalent of the vinclozolin acceptable daily intake (ADI) used for consumer regulatory purposes. Even the highest concentrations were, however, at least 10 times below the no observable adverse effect level (NOAEL) based on animal studies. Analysis of physical examination and laboratory data provided no evidence of hormonal responses induced by vinclozolin. Furthermore, no evidence of liver injury, prostate changes, cataract formation, or haemolytic anaemia was found. CONCLUSION: There was no evidence of any health effects induced by vinclozolin among employees with potential long term exposure. In particular, no antiandrogenic effects were found.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1128201/

Record 1635

pmcid: PMC2541805

pmid: 7888856

doi: 10.1136/bmj.309.6964.1292

title: ABC of breast diseases. Breast cancer: treatment of elderly patients and uncommon conditions.

authors

- Dixon JM
- Sainsbury JR
- Rodger A

journal: BMJ (Clinical research ed.)

journalAbbr: BMJ

year: 1994

pubdate: 1994 Nov 12

volume: 309

issue: 6964

pages: 1292-5

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2541805/

Record 1636

pmcid: PMC2541002

pmid: 7950574

doi: 10.1136/bmj.309.6957.797

title: ABC of breast diseases. Congenital problems and aberrations of normal breast development and involution.

authors

- Dixon JM
- Mansel RE

journal: BMJ (Clinical research ed.)

journalAbbr: BMJ

year: 1994

pubdate: 1994 Sep 24

volume: 309

issue: 6957

pages: 797-800

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2541002/

Record 1637

pmcid: PMC1317760

pmid: 16558265

doi:

title: Anabolic steroid use in the adolescent athlete.

authors

- Potteiger JA
- Stilger VG

journal: Journal of athletic training

journalAbbr: J Athl Train

year: 1994

pubdate: 1994 Mar

volume: 29

issue: 1

pages: 60-4

abstract: Recent surveys indicate that the use of androgenic-anabolic steroids (anabolic steroids) is prevalent among adolescent athletes, particularly those in high school. The cost of clinical drug testing makes it impractical to use random testing to identify users of these ergogenic aids. The athletic trainer is often in a position to identify anabolic steroid users if he/she knows the clinical signs and symptoms. In this article, we briefly discuss the history of anabolic steroid use, how they work, their potentially dangerous side effects, evidence of increased use by adolescent athletes, and a list of clinical signs and symptoms. Finally, we suggest strategies that may help athletic trainers counsel athletes about anabolic steroid use.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1317760/

Record 1638

pmcid: PMC2397830

pmid: 7824423

doi: 10.1136/pgmj.70.829.836

title: Endocrine assessment of impotence--pitfalls of measuring serum testosterone without sex-hormone-binding globulin.

authors

- Hardy KJ
- Seckl JR

journal: Postgraduate medical journal

journalAbbr: Postgrad Med J

year: 1994

pubdate: 1994 Nov

volume: 70

issue: 829

pages: 836-7

abstract: The pitfalls of measuring only total serum testosterone are illustrated by a 52 year old man whose hyperprolactinaemia was associated with normal total serum testosterone but a raised sex-hormone-binding globulin, giving a low free testosterone. Prolactin suppression with bromocriptine normalized sex-hormone-binding globulin and free testosterone, and restored potency and energy after 30 years of impotence and tiredness.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2397830/

Record 1639

pmcid: PMC2542783

pmid: 8136667

doi: 10.1136/bmj.308.6927.503

title: Risk of gynaecomastia associated with cimetidine, omeprazole, and other antiulcer drugs.

authors

- García Rodríguez LA
- Jick H

journal: BMJ (Clinical research ed.)

journalAbbr: BMJ

year: 1994

pubdate: 1994 Feb 19

volume: 308

issue: 6927

pages: 503-6

abstract: OBJECTIVE: To study the risk of gynaecomastia associated with cimetidine, misoprostol, omeprazole and ranitidine. DESIGN: Open cohort study with nested case-control analysis. SETTING: General practices in United Kingdom that had computerised offices, 1989-92. SUBJECTS: 81,535 men aged 25-84 years who received at least one prescription for cimetidine, misoprostol, omeprazole, or ranitidine during the study period. MAIN OUTCOME MEASURES: New occurrences of idiopathic gynaecomastia diagnosed by general practitioner. RESULTS: The relative risk of gynaecomastia for current users of cimetidine compared with non-users was 7.2 (95% confidence interval 4.5 to 11.3). Relative risks for misoprostol, omeprazole, and ranitidine were 2.0 (0.1 to 10.7), 0.6 (0.1 to 3.3), and 1.5 (0.8 to 2.6), respectively. Current users of cimetidine on a daily dose > or = 1000 mg had more than 40 times the risk of developing gynaecomastia than non-users. The period of highest risk was seven to 12 months after starting cimetidine treatment. Spironolactone (relative risk 9.3 (3.3 to 26.1)) and verapamil (9.7 (2.6 to 36.0)) were associated with a relative risk of gynaecomastia comparable to one for cimetidine. CONCLUSIONS: Use of cimetidine, but not the three other antiulcer drugs, is associated with a substantially greater risk of gynaecomastia in men. A strong dose-response relation was present among cimetidine users.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2542783/

Record 1640

pmcid: PMC2399848

pmid: 8208649

doi: 10.1136/pgmj.69.812.481

title: A feminizing adrenocortical carcinoma presenting with gynaecomastia.

authors

- Lanigan D
- Choa RG
- Evans J

journal: Postgraduate medical journal

journalAbbr: Postgrad Med J

year: 1993

pubdate: 1993 Jun

volume: 69

issue: 812

pages: 481-3

abstract: A case is presented in which gynaecomastia was the sole initial presenting symptom of a feminizing adrenocortical carcinoma. This rare pathological lesion is discussed.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2399848/

Record 1641

pmcid: PMC1519930

pmid: 8243404

doi: 10.1289/ehp.93101s239

title: Environmental factors influencing growth and pubertal development.

authors

- Delemarre-van de Waal HA

journal: Environmental health perspectives

journalAbbr: Environ Health Perspect

year: 1993

pubdate: 1993 Jul

volume: 101 Suppl 2

issue: Suppl 2

pages: 39-44

abstract: Postnatal growth is based on hereditary signals and environmental factors in a complex regulatory network. Each factor must be in an optimal state for normal growth of the child. Fetal conditions may also have consequences on postnatal height. Intrauterine growth retardation can be recovered postnatally, although postnatal growth remains depressed in about one-third of cases. After birth, the environment may exert either a positive or negative effect on growth. In underdeveloped countries, malnutrition plays a major role in inhibiting the growth process. Children from families of higher socioeconomic classes are taller than their coevals in the lower socioeconomic groups. Urbanization also has a positive effect on growth. Better child care is supported by sufficient food supply, appropriate health and sanitation services, and a higher level of education. Over the last century, these factors have induced a taller stature and a more rapid maturity in Europe, North America, and Australia; a phenomenon which has been referred to as "the secular trend" in growth. Recently, a secular trend has also been reported in some developing countries. Although urbanization in general appears to be associated with better conditions of living, this is not the case in the slums of South America or in Africa where rural children are better off than children living in the poor cities. This paper describes in more detail the different hereditary and environmental factors that act during the fetal period and postnatally, and which play a role in human growth and pubertal development.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1519930/

Record 1642

pmcid: PMC2980987

pmid: 21776101

doi:

title: Dysmorphophobia with bipolar affective disorder : a case report.

authors

- Sengupta SN
- Das P
- Kamble MV
- Saxena S

journal: Indian journal of psychiatry

journalAbbr: Indian J Psychiatry

year: 1992

pubdate: 1992 Jan

volume: 34

issue: 1

pages: 57-8

abstract: This case report describes the progression of dysmorphophobic symptoms in the dimension of intensity from a normal age related concern, to primary dysmorpliophobia, to a somatic delusion during depressive and manic phases of a bipolar affective disorder. The depressive episodes were resistant to drug treatment but responded to ECTs.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2980987/

Record 1643

pmcid: PMC1882563

pmid: 1392958

doi: 10.1136/bmj.305.6851.451

title: Endocrine adverse effects of omeprazole.

authors

- Lindquist M
- Edwards IR

journal: BMJ (Clinical research ed.)

journalAbbr: BMJ

year: 1992

pubdate: 1992 Aug 22

volume: 305

issue: 6851

pages: 451-2

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1882563/

Record 1644

pmcid: PMC1682625

pmid: 1347967

doi:

title: Maternal uniparental isodisomy of chromosome 14: association with autosomal recessive rod monochromacy.

authors

- Pentao L
- Lewis RA
- Ledbetter DH
- Patel PI
- Lupski JR

journal: American journal of human genetics

journalAbbr: Am J Hum Genet

year: 1992

pubdate: 1992 Apr

volume: 50

issue: 4

pages: 690-9

abstract: Rod monochromacy (complete congenital achromatopsia) is inherited as an autosomal recessive trait of unknown genetic location. The disorder is characterized by total absence of color discrimination because retinal cone photoreceptors do not develop; systemic features do not occur. A 20-year-old white female with rod monochromacy presented with short stature (less than 5th percentile), mild developmental delay, premature puberty, small hands and feet (length less than 5th percentile), minimal dysmorphism, and a reproductive history of three consecutive first-trimester miscarriages. Cytogenetic analysis showed 45,XX,rob(14;14) in all 30 cells examined. Southern analysis of DNA from the patient and her phenotypically normal mother and two brothers (her father is deceased) ascertained the parental origin of the 14;14 Robertsonian translocation. Analysis of RFLPs associated with nine VNTR probes and two dinucleotide repeat polymorphisms from chromosome 14 demonstrated that the patient had inherited two copies of a single allele, each of which was maternally derived. A fully informative RFLP analysis of three probes from chromosome 14 enabled reconstruction of the paternal haplotype and showed the lack of any paternal contribution to the subject. These data are consistent with maternal isodisomy for all portions of chromosome 14 tested by these markers. This finding suggests that rod monochromacy maps to chromosome 14, and it emphasizes the importance of uniparental isodisomy to provide a putative chromosomal assignment of a gene for a rare autosomal recessive disorder.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1682625/

Record 1645

pmcid: PMC1683077

pmid:

doi:

title: Poster presentations.

authors

journal: American Journal of Human Genetics

journalAbbr: Am J Hum Genet

year: 1992

pubdate: 1992 Oct

volume: 51

issue: Suppl

pages: A56-A119

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1683077/

Record 1646

pmcid: PMC1403614

pmid:

doi:

title: PROGRAMS, PRACTICES, PEOPLE.

authors

journal: Public Health Reports

journalAbbr: Public Health Rep

year: 1992

pubdate: 1992 Jan-Feb

volume: 107

issue: 1

pages: 121-7

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1403614/

Record 1647

pmcid: PMC2499328

pmid: 1996868

doi:

title: A comparison of danazol and placebo in the treatment of adult idiopathic gynaecomastia: results of a prospective study in 55 patients.

authors

- Galea MH
- Blamey RW

journal: Annals of the Royal College of Surgeons of England

journalAbbr: Ann R Coll Surg Engl

year: 1991

pubdate: 1991 Jan

volume: 73

issue: 1

pages: 65

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2499328/

Record 1648

pmcid: PMC1792847

pmid: 2001118

doi: 10.1136/adc.66.2.263

title: Accidental poisoning in children: can we admit fewer children with safety?

authors

- Sibert JR
- Routledge PA

journal: Archives of disease in childhood

journalAbbr: Arch Dis Child

year: 1991

pubdate: 1991 Feb

volume: 66

issue: 2

pages: 263-6

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1792847/

Record 1649

pmcid: PMC1188278

pmid:

doi:

title: Administrative content — journal masthead, notices, indexes, etc.

authors

journal: J Psychiatry Neurosci

journalAbbr: J Psychiatry Neurosci

year: 1991

pubdate: 1991 Mar

volume: 16

issue: 1

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1188278/

Record 1650

pmcid: PMC3065017

pmid: 21451731

doi: 10.1080/00224499109551619

title: Body Image for Women: Conceptualization, Assessment, and a Test of its Importance to Sexual Dysfunction and Medical Illness.

authors

- Andersen BL
- Legrand J

journal: Journal of sex research

journalAbbr: J Sex Res

year: 1991

pubdate: 1991

volume: 28

issue: 3

pages: 457-477

abstract: The content and valence of women's body image attitudes, general and enduring positive or negative feelings about the body, are studied with psychometric analyses of measures and contrasted groups. Data from two frequently used measures (Body Image Scale, Derogatis & Melisaratos, 1979; Body Satisfaction Scale, Berscheid, Walster & Bohrnstedt, 1973) provided an evaluation of the construct and the assessment of body image. Two studies are provided. The construct analyses suggest two contents for body attitude measures: a general factor of body, facial, and sexual (genital and breast) items, and a second factor assessing weight and/or its body correlates-the hips, thighs, and buttocks. Also, a method factor, a response style of negativity, may be important. Body image attitudes are correlated with some conceptually relevant criteria, such as interest in engaging in sexual activity; however, these relationships do not appear sufficiently strong to predict behavior, such as the occurrence or resolution of sexual dysfunction. Generalized body image disturbance as currently conceptualized and assessed may be difficult to document, particularly when item content and response styles are considered.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC3065017/

Record 1651

pmcid: PMC1004492

pmid: 1888201

doi: 10.1136/ard.50.8.577

title: Breast vasculitis in association with breast gigantism in a pregnant patient with systemic lupus erythematosus.

authors

- Propper DJ
- Reid DM
- Stankler L
- Eastmond CJ

journal: Annals of the rheumatic diseases

journalAbbr: Ann Rheum Dis

year: 1991

pubdate: 1991 Aug

volume: 50

issue: 8

pages: 577-8

abstract: A 24 year old woman with systemic lupus erythematosus (SLE) developed widespread necrotic skin ulceration and gigantism of both breasts during an exacerbation of SLE in the last trimester of her second pregnancy. Over the remainder of the pregnancy the ulceration was only controlled by high dose corticosteroids. After parturition, however, it was possible to reduce the steroid dose without recurrence of the ulceration.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1004492/

Record 1652

pmcid: PMC1668949

pmid:

doi:

title: Letter from Chicago: Joe Doe's new year.

authors

- Dunea G

journal: BMJ : British Medical Journal

journalAbbr: BMJ

year: 1991

pubdate: 1991 Feb 2

volume: 302

issue: 6771

pages: 283-4

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1668949/

Record 1653

pmcid: PMC2145662

pmid: 21229080

doi:

title: Prepubertal Gynecomastia: A cause for concern.

authors

- Bass J
- Sochett E

journal: Canadian family physician Medecin de famille canadien

journalAbbr: Can Fam Physician

year: 1991

pubdate: 1991 Apr

volume: 37

issue:

pages: 995-1026

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2145662/

Record 1654

pmcid: PMC1335872

pmid: 1751934

doi:

title: Safety of polyurethane-covered breast implants. Expert Panel on the Safety of Polyurethane-covered Breast Implants.

authors

journal: CMAJ : Canadian Medical Association journal = journal de l'Association medicale canadienne

journalAbbr: CMAJ

year: 1991

pubdate: 1991 Nov 1

volume: 145

issue: 9

pages: 1125-32

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1335872/

Record 1655

pmcid: PMC1670075

pmid: 1676318

doi: 10.1136/bmj.302.6789.1367

title: Single dose cabergoline versus bromocriptine in inhibition of puerperal lactation: randomised, double blind, multicentre study. European Multicentre Study Group for Cabergoline in Lactation Inhibition.

authors

journal: BMJ (Clinical research ed.)

journalAbbr: BMJ

year: 1991

pubdate: 1991 Jun 8

volume: 302

issue: 6789

pages: 1367-71

abstract: OBJECTIVE: To compare the efficacy and safety of a single dose of 1 mg of cabergoline with that of bromocriptine 2.5 mg twice daily for 14 days in the inhibition of puerperal lactation. DESIGN: Prospective, randomised, double blind, parallel group, multicentre study. SETTING: University of hospital departments of obstetrics and gynaecology in different European countries. SUBJECTS: 272 puerperal women not wishing to lactate (136 randomised to each drug). INTERVENTIONS: Women randomised to cabergoline received two 0.5 mg tablets of cabergoline and one placebo tablet within 27 hours after delivery and then placebo twice daily for 14 days. Those randomised to bromocriptine received 2.5 mg of bromocriptine and two placebo tablets within 27 hours and then 2.5 mg of bromocriptine twice daily for 14 days. MAIN OUTCOME MEASURES: Success of treatment (complete or partial) according to milk secretion, breast engorgement, and breast pain; rebound symptomatology; serum prolactin concentrations; and number of adverse events. RESULTS: Complete success was achieved in 106 of 136 women randomised to cabergoline and in 94 of 136 randomised to bromocriptine and partial success in 21 and 33 women respectively. Rebound breast symptomatology occurred respectively in five and 23 women with complete success up to day 15 (p less than 0.0001). Serum prolactin concentrations dropped considerably with both drugs from day 2 to day 15; a prolactin secretion rebound effect was observed in women treated with bromocriptine. cabergoline and 36 receiving bromocriptine (p = 0.054), occurring most during the first treatment day. CONCLUSION: A single 1 mg dose of cabergoline is at least as effective as bromocriptine 2.5 mg twice daily for 14 days in preventing puerperal lactation. Because of the considerably lower rate of rebound breast activity and adverse events and the simpler administration schedule cabergoline should be the drug of choice for lactation inhibition.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1670075/

Record 1656

pmcid: PMC2145637

pmid: 21229077

doi:

title: When Breast-feeding is not Contraindicated: Do you know when to stop breast-feeding?

authors

- Newman J

journal: Canadian family physician Medecin de famille canadien

journalAbbr: Can Fam Physician

year: 1991

pubdate: 1991 Apr

volume: 37

issue:

pages: 969-75

abstract: As more mothers elect to breast-feed, more concomitant problems in mothers and babies are reported that are thought to contraindicate breast-feeding. Many frequently cited maternal and infant reasons for stopping breast-feeding are not valid. Breast-feeding can usually be maintained if the physician remembers that breast-feeding is important for the baby and mother and not simply another feeding method.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2145637/

Record 1657

pmcid: PMC1017244

pmid: 2246774

doi: 10.1136/jmg.27.10.637

title: A de novo interstitial deletion of 15(q21.2q22.1) in a moderately retarded adult male.

authors

- Martin F
- Platt J
- Tawn EJ
- Burn J

journal: Journal of medical genetics

journalAbbr: J Med Genet

year: 1990

pubdate: 1990 Oct

volume: 27

issue: 10

pages: 637-9

abstract: An adult male is described with a de novo deletion 15q21.2-q22.1. He shares some minor dysmorphic features with similar cases but the degree of mental retardation is markedly less severe.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1017244/

Record 1658

pmcid: PMC1792429

pmid: 2386381

doi: 10.1136/adc.65.7.760

title: Growth after gut resection for Crohn's disease.

authors

- McLain BI
- Davidson PM
- Stokes KB
- Beasley SW

journal: Archives of disease in childhood

journalAbbr: Arch Dis Child

year: 1990

pubdate: 1990 Jul

volume: 65

issue: 7

pages: 760-2

abstract: Seventeen children underwent 19 bowel resections as part of their management of Crohn's disease. Thirteen children had evidence of retardation of linear growth preoperatively, of whom 12 exhibited catch up growth, crossing at least one centile band, after resection. Seven of the 12 showed early signs of puberty at the time of resection and one child was regarded as being fully pubertal. All children were symptomatic before their surgery; at one year 11 were asymptomatic and three others were substantially improved. Surgical resection of localised Crohn's disease, followed by adequate nutritional support has lead to catch up growth and a prolonged symptom free period in most of our patients and this was not limited by their age or stage of puberty.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1792429/

Record 1659

pmcid: PMC1662310

pmid: 2107949

doi: 10.1136/bmj.300.6723.541

title: Gynaecomastia induced by angiotensin converting enzyme inhibitor.

authors

- Nakamura Y
- Yoshimoto K
- Saima S

journal: BMJ (Clinical research ed.)

journalAbbr: BMJ

year: 1990

pubdate: 1990 Feb 24

volume: 300

issue: 6723

pages: 541

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1662310/

Record 1660

pmcid: PMC2280134

pmid: 21233923

doi:

title: Problem-Solving Formula for Failure to Thrive in Breast-fed Infants.

authors

- Livingstone VH

journal: Canadian family physician Medecin de famille canadien

journalAbbr: Can Fam Physician

year: 1990

pubdate: 1990 Sep

volume: 36

issue:

pages: 1541-5

abstract: Failure to thrive and insufficient milk are common reasons given for terminating breast-feeding early. Mothers often doubt their natural ability to successfully suckle a young infant. This perceived inadequacy may be reinforced by health professionals who advise supplemental formula feeds to ensure rapid weight gain before an accurate diagnosis has been made. The growth of healthy infants depends on maternal milk production, milk transfer at the breasts, and the quantity and quality of milk intake by the baby. Problems with any one of these areas can present as failure to thrive. The author focuses on a diagnostic approach to failure to thrive in a breast-fed infant and outlines ways to assess maternal lactation ability, milk production, milk transfer, and milk intake. The diagnosis of failure to thrive is often simple and follows sound physiological and anatomical principles.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2280134/

Record 1661

pmcid: PMC1268604

pmid: 2910406

doi:

title: Canada's doctors--who they are and what they do: lessons from the CMA's 1986 manpower survey.

authors

- Adams O

journal: CMAJ : Canadian Medical Association journal = journal de l'Association medicale canadienne

journalAbbr: CMAJ

year: 1989

pubdate: 1989 Jan 15

volume: 140

issue: 2

pages: 212-21

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1268604/

Record 1662

pmcid: PMC1837759

pmid: 2510882

doi: 10.1136/bmj.299.6704.899

title: Colposcopy services in the West Midlands region.

authors

- Woodman CB
- Jordan JA

journal: BMJ (Clinical research ed.)

journalAbbr: BMJ

year: 1989

pubdate: 1989 Oct 7

volume: 299

issue: 6704

pages: 899-901

abstract: A survey of all 72 consultant gynaecologists in the West Midlands region was carried out to determine their views on colposcopy services. All districts provided a colposcopy service and 47 consultants practised colposcopy. The consultants differed considerably in their views on criteria for referring women for investigation after smears. All but one thought that a positive smear result was an indication for immediate referral but whereas 55 thought that women with one or more inflammatory smear should be referred, 17 did not believe this to be necessary. Sixty seven consultants thought a preoperative colposcopic assessment desirable, but 10 had to ration referrals because of limited resources. Four forms of treatment were used: 17 districts had a carbon dioxide laser, eight used low voltage diathermy loop excision, two had only a cold coagulator, and one used only cold cautery. There was widespread dissatisfaction with resources and about the increasing amount of time spent on the service at the expense of other work. Consultants were also concerned about the relevance of much of what was done to the aim of the screening programme--reducing mortality from cervical cancer. Although additional resources are needed in some districts, referral policies, indications for colposcopic assessment, training, choice of treatment, and the value of follow up need to be reviewed if the service is to be improved.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1837759/

Record 1663

pmcid: PMC2719966

pmid: 2600238

doi: 10.1037//0022-006x.57.6.683

title: Controlled prospective longitudinal study of women with cancer: I. Sexual functioning outcomes.

authors

- Andersen BL
- Anderson B
- deProsse C

journal: Journal of consulting and clinical psychology

journalAbbr: J Consult Clin Psychol

year: 1989

pubdate: 1989 Dec

volume: 57

issue: 6

pages: 683-91

abstract: The incidence and etiology of sexual difficulties for women with survivable cancer were studied. Women with early stage gynecologic cancer (n = 47) were assessed after their diagnosis but prior to treatment and then reassessed at 4, 8, and 12 months posttreatment. Sexual and medical outcomes were compared with data from members of two matched comparison groups who were also assessed longitudinally: women diagnosed and treated for benign gynecologic disease (n = 18) and gynecologically healthy women (n = 57). Global sexual behavior disruption did not occur, but the frequency of intercourse declined for women treated for disease, whether malignant or benign. In relation to the sexual response cycle, diminution of sexual excitement is pronounced for women with disease; however, this difficulty is more severe and distressing for women with cancer, possibly due to significant coital and postcoital pain, premature menopause, treatment side effects, or a combination. Changes in desire, orgasm, and resolution phases of the sexual response cycle may also occur, but they are of lesser magnitude or duration or both. Approximately 30% of the women treated for cancer were diagnosed with a sexual dysfunction. The nature, early timing, and maintenance of sexual functioning morbidity suggest the instrumental role that cancer and cancer treatments play in these deficits (particularly arousal problems) and suggest that preventive therapies are necessary.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2719966/

Record 1664

pmcid: PMC1009778

pmid: 2751933

doi: 10.1136/oem.46.5.341

title: Occupational exposure to zeranol, an animal growth promoter.

authors

- Aw TC
- Smith AB
- Stephenson RL
- Glueck CJ

journal: British journal of industrial medicine

journalAbbr: Br J Ind Med

year: 1989

pubdate: 1989 May

volume: 46

issue: 5

pages: 341-6

abstract: Zeranol (3,4,5,6,7,8,9,10,11,12-Decahydro-7,14,16-trihydroxy-3-methyl-1H-2 -benzoxacyclotetradecin-1-one) is a synthetic oestrogenic agent used as an animal growth promoter. The effects of occupational exposure to zeranol in 11 exposed workers from a pelletising plant and 14 nonexposed subjects were assessed. A questionnaire showed that more breast symptoms were reported by male and female plant workers compared with non-exposed subjects, although the difference was not statistically significant. Clinical

assessment showed no cases of gynaecomastia in all the male participants. Blood samples analysed by high performance liquid chromatography for zeranol, its precursor zearalenone, and its main metabolites did not show any of these compounds above the laboratory limit of detection. Serum levels of follicle stimulating hormone (FSH), luteinising hormone (LH), prolactin, and oestradiol showed no striking differences between the exposed and the non-exposed subjects. Total and low density lipoprotein cholesterol (LDL cholesterol) levels did not significantly differ between the two groups but mean high density lipoprotein cholesterol (HDL cholesterol) levels were higher in the exposed group; this could be due to relatively high HDL cholesterol in two women exposed to zeranol or relatively low HDL cholesterol in three non-exposed men.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1009778/

Record 1665

pmcid: PMC1451365

pmid: 2776087

doi:

title: Pacific rim publishing piracy.

authors

- Hegele RA

journal: CMAJ : Canadian Medical Association journal = journal de l'Association medicale canadienne

journalAbbr: CMAJ

year: 1989

pubdate: 1989 Sep 15

volume: 141

issue: 6

pages: 512-3

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1451365/

Record 1666

pmcid: PMC2280178

pmid: 20469507

doi:

title: Psychotropic drugs in children: a guide for the family physician.

authors

- Goldberg B

journal: Canadian family physician Medecin de famille canadien

journalAbbr: Can Fam Physician

year: 1989

pubdate: 1989 Dec

volume: 35

issue:

pages: 2459-63

abstract: Pediatric psychopharmacology, an emerging field, is a field fraught with potential abuses, about which

consumer groups are concerned. These concerns have initiated legislation in some jurisdictions of Canada. Abuse is most likely to occur in the absence of other professional services, such as psychology, education, or social services. After careful assessment, diagnosis, and consultation, if necessary, the family physician can prescribe these medications responsibly, sharing the decision-making with the child and parent.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2280178/

Record 1667

pmcid: PMC1451446

pmid: 2804855

doi:

title: Reflections from the trenches.

authors

- McCann D

journal: CMAJ : Canadian Medical Association journal = journal de l'Association medicale canadienne

journalAbbr: CMAJ

year: 1989

pubdate: 1989 Nov 1

volume: 141

issue: 9

pages: 968-9

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1451446/

Record 1668

pmcid: PMC1451340

pmid: 2629722

doi:

title: Standards of beauty: Degas v. Kirov.

authors

- Wolfish MG

journal: CMAJ : Canadian Medical Association journal = journal de l'Association medicale canadienne

journalAbbr: CMAJ

year: 1989

pubdate: 1989 Sep 15

volume: 141

issue: 6

pages: 513

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1451340/

Record 1669

pmcid: PMC2498995

pmid: 2774450

doi:

title: Surgical treatment of grade III gynaecomastia.

authors

- Ward CM
- Khalid K

journal: Annals of the Royal College of Surgeons of England

journalAbbr: Ann R Coll Surg Engl

year: 1989

pubdate: 1989 Jul

volume: 71

issue: 4

pages: 226-8

abstract: Gross enlargement of the male breast with skin redundancy and ptosis cannot be effectively managed by subcutaneous mastectomy alone. A technique is described in which the excess breast tissue and skin are excised along a design which allows the nipple-areolar complex, mounted on a vertical deepithelialised pedicle, to lie in a natural position. A horizontal scar is an unavoidable exchange for this particular operation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2498995/

Record 1670

pmcid: PMC1451320

pmid:

doi:

title: Third World aid.

authors

- Godfrey CM

journal: CMAJ: Canadian Medical Association Journal

journalAbbr: CMAJ

year: 1989

pubdate: 1989 Oct 15

volume: 141

issue: 8

pages: 768-9

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1451320/

Record 1671

pmcid: PMC1451291

pmid:

doi:

title: Third World aid.

authors

- McCallum W

journal: CMAJ: Canadian Medical Association Journal

journalAbbr: CMAJ

year: 1989

pubdate: 1989 Oct 15

volume: 141

issue: 8

pages: 769-70

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1451291/

Record 1672

pmcid: PMC2428676

pmid: 3200782

doi: 10.1136/pgmj.64.751.386

title: Alveolar soft-part sarcoma: a hormone-sensitive tumour?

authors

- Pang JA
- Yeung TF
- Cockram CS

journal: Postgraduate medical journal

journalAbbr: Postgrad Med J

year: 1988

pubdate: 1988 May

volume: 64

issue: 751

pages: 386-8

abstract: Alveolar soft-part sarcoma is a rare, slow-growing, malignant tumour which metastasizes frequently to the lungs. Treatment with radiotherapy or chemotherapy is largely ineffective, and most patients die within 4 years of developing metastases. We report an unusual patient who survived 9 years after detection of pulmonary metastases. Initial treatment with cytotoxic chemotherapy was ineffective. However, during subsequent treatment with Chinese herbs when she developed galactorrhoea and amenorrhoea (probably due to drug-induced hyperprolactinaemia), her lung metastases regressed, and it is likely that this contributed to her prolonged survival. We hypothesize that alveolar soft-part sarcoma may be a hormone-sensitive tumour, and hormonal manipulation may be an alternative form of treatment which is worth considering.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2428676/

Record 1673

pmcid: PMC1715419

pmid:

doi:

title: Clinical genetics.

authors

journal: American Journal of Human Genetics

journalAbbr: Am J Hum Genet

year: 1988

pubdate: 1988 Sep

volume: 43

issue: Suppl

pages: A36-A100

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1715419/

Record 1674

pmcid: PMC2545690

pmid: 3133043

doi: 10.1136/bmj.296.6631.1262-c

title: Gynaecomastia associated with captopril.

authors

- Markusse HM
- Meyboom RH

journal: British medical journal (Clinical research ed.)

journalAbbr: Br Med J (Clin Res Ed)

year: 1988

pubdate: 1988 Apr 30

volume: 296

issue: 6631

pages: 1262-3

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2545690/

Record 1675

pmcid: PMC1002107

pmid: 3555359

doi: 10.1136/ard.46.3.237

title: A controlled trial of nandrolone decanoate in the treatment of rheumatoid arthritis in postmenopausal women.

authors

- Bird HA
- Burkinshaw L
- Pearson D

- Atkinson PJ
- Leatham PA
- Hill J
- Raven A
- Wright V

journal: Annals of the rheumatic diseases

journalAbbr: Ann Rheum Dis

year: 1987

pubdate: 1987 Mar

volume: 46

issue: 3

pages: 237-43

abstract: To determine whether an anabolic steroid had any benefit in the treatment of rheumatoid arthritis 47 patients entered a parallel group study. Twenty four received nandrolone decanoate 50 mg intramuscularly every third week for two years and 23 patients received no anabolic steroids. Other therapy was unaltered. Patients attended for clinical and biochemical assessments as well as the objective assessments of elementary body composition by in vivo neutron activation analysis and measurement of the mineral content of the distal femur by single photon absorptiometry on five occasions. A modest clinical deterioration (except for grip strength) was seen in both groups. No significant changes in calcium or alkaline phosphatase were seen. There was no significant change in total body calcium, total body phosphorus, body weight, or bone index/bone width measurements in either group. Significant increases occurred in total body nitrogen, total body potassium, haemoglobin, and packed cell volume (by six months) in the group treated with nandrolone decanoate. Comparison of 10 patients in the group treated with nandrolone decanoate also receiving oral steroid therapy with 14 patients in this group not receiving oral steroid therapy showed no significant differences. The main side effect of nandrolone decanoate was hoarseness. No radiological changes were seen. Nandrolone decanoate, in a dose that produces a significant anabolic effect, has no demonstrable action on bone metabolism in rheumatoid arthritis but may improve the chronic anaemia by six months.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1002107/

Record 1676

pmcid: PMC1267475

pmid: 3676974

doi:

title: Cosmetic surgery is coming out of the closet.

authors

- Trent B

journal: CMAJ : Canadian Medical Association journal = journal de l'Association medicale canadienne

journalAbbr: CMAJ

year: 1987

pubdate: 1987 Dec 15

volume: 137

issue: 12

pages: 1123-4

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1267475/

Record 1677

pmcid: PMC2218495

pmid:

doi:

title: Guide to Anesthesia.

authors

- Houston S

journal: Canadian Family Physician

journalAbbr: Can Fam Physician

year: 1987

pubdate: 1987 May

volume: 33

issue:

pages: 1334-5

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2218495/

Record 1678

pmcid: PMC1245652

pmid: 3103827

doi: 10.1136/bmj.294.6572.612

title: Gynaecomastia induced by phenytoin in men with epilepsy.

authors

- Monson JP
- Scott DF

journal: British medical journal (Clinical research ed.)

journalAbbr: Br Med J (Clin Res Ed)

year: 1987

pubdate: 1987 Mar 7

volume: 294

issue: 6572

pages: 612

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1245652/

Record 1679

pmcid: PMC1492786

pmid:

doi:

title: Indoor air quality subject of Montreal conference.

authors

- Trainor D

journal: CMAJ: Canadian Medical Association Journal

journalAbbr: CMAJ

year: 1987

pubdate: 1987 Sep 1

volume: 137

issue: 5

pages: 435-6

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1492786/

Record 1680

pmcid: PMC1492562

pmid:

doi:

title: Jake Epp: The Teflon Tory.

authors

- Gray C

journal: CMAJ: Canadian Medical Association Journal

journalAbbr: CMAJ

year: 1987

pubdate: 1987 May 15

volume: 136

issue: 10

pages: 1102-3

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1492562/

Record 1681

pmcid: PMC1477879

pmid:

doi:

title: PROGRAMS, PRACTICES, PEOPLE.

authors

journal: Public Health Reports

journalAbbr: Public Health Rep

year: 1987

pubdate: 1987 Jul-Aug

volume: 102

issue: 4

pages: 452-8

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1477879/

Record 1682

pmcid: PMC1267359

pmid: 3327580

doi:

title: When doctors went to war.

authors

- Bourne M

journal: CMAJ : Canadian Medical Association journal = journal de l'Association medicale canadienne

journalAbbr: CMAJ

year: 1987

pubdate: 1987 Nov 1

volume: 137

issue: 9

pages: 834-6

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1267359/

Record 1683

pmcid: PMC1339411

pmid:

doi:

title: Cover page.

authors

journal: Br Med J (Clin Res Ed)

journalAbbr: Br Med J (Clin Res Ed)

year: 1986

pubdate: 1986 Feb 15

volume: 292

issue: 6518

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1339411/

Record 1684

pmcid: PMC1491404

pmid: 3779562

doi:

title: Doctors and torture: a tortuous issue.

authors

- Gilmore A

journal: CMAJ : Canadian Medical Association journal = journal de l'Association medicale canadienne

journalAbbr: CMAJ

year: 1986

pubdate: 1986 Dec 1

volume: 135

issue: 11

pages: 1305-8

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1491404/

Record 1685

pmcid: PMC1339425

pmid: 3081118

doi: 10.1136/bmj.292.6518.446

title: Heart failure and breast enlargement suggesting cancer.

authors

- McElligott G
- Harrington MG

journal: British medical journal (Clinical research ed.)

journalAbbr: Br Med J (Clin Res Ed)

year: 1986

pubdate: 1986 Feb 15

volume: 292

issue: 6518

pages: 446

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1339425/

Record 1686

pmcid: PMC1339846

pmid:

doi:

title: Original table of contents page.

authors

journal: Br Med J (Clin Res Ed)

journalAbbr: Br Med J (Clin Res Ed)

year: 1986

pubdate: 1986 Apr 5

volume: 292

issue: 6525

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1339846/

Record 1687

pmcid: PMC1339412

pmid:

doi:

title: Original table of contents page.

authors

journal: Br Med J (Clin Res Ed)

journalAbbr: Br Med J (Clin Res Ed)

year: 1986

pubdate: 1986 Feb 15

volume: 292

issue: 6518

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1339412/

Record 1688

pmcid: PMC1339960

pmid:

doi:

title: Original table of contents page.

authors

journal: Br Med J (Clin Res Ed)

journalAbbr: Br Med J (Clin Res Ed)

year: 1986

pubdate: 1986 Mar 29

volume: 292

issue: 6524

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1339960/

Record 1689

pmcid: PMC1339768

pmid:

doi:

title: Original table of contents page.

authors

journal: Br Med J (Clin Res Ed)

journalAbbr: Br Med J (Clin Res Ed)

year: 1986

pubdate: 1986 Mar 15

volume: 292

issue: 6522

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1339768/

Record 1690

pmcid: PMC1340019

pmid:

doi:

title: Points: Heart failure and breast enlargement suggesting cancer.

authors

- Green ST

journal: British Medical Journal (Clinical research ed.)

journalAbbr: Br Med J (Clin Res Ed)

year: 1986

pubdate: 1986 Mar 29

volume: 292

issue: 6524

pages: 903

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1340019/

Record 1691

pmcid: PMC1339895

pmid:

doi:

title: Points: Heart failure and breast enlargement suggesting cancer.

authors

- Needham D
- Green ST

journal: British Medical Journal (Clinical research ed.)

journalAbbr: Br Med J (Clin Res Ed)

year: 1986

pubdate: 1986 Apr 5

volume: 292

issue: 6525

pages: 961

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1339895/

Record 1692

pmcid: PMC1339833

pmid:

doi:

title: Points: Heart failure and breast enlargement suggesting cancer.

authors

- Murray WJG
- Kiff PS

journal: British Medical Journal (Clinical research ed.)

journalAbbr: Br Med J (Clin Res Ed)

year: 1986

pubdate: 1986 Mar 15

volume: 292

issue: 6522

pages: 766

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1339833/

Record 1693

pmcid: PMC2705956

pmid: 3955525

doi: 10.1002/1097-0142(19860501)57:9<1880::aid-cncr2820570930>3.0.co;2-v

title: Sexual dysfunction and signs of gynecologic cancer.

authors

- Andersen BL
- Lachenbruch PA
- Anderson B

- deProsse C

  journal: Cancer

  journalAbbr: Cancer

  year: 1986

  pubdate: 1986 May 1

  volume: 57

  issue: 9

  pages: 1880-6

  abstract: Forty-one women recently diagnosed with early-stage cervical or endometrial cancer and a matched group of healthy women in no gynecologic distress, participated in a detailed assessment of their sexual functioning. Data included the range and frequency of sexual behavior, level of sexual responsiveness, and the presence of sexual dysfunction. Multivariate analyses of variance indicated that prior to the onset of cancer signs/symptoms the gynecologic cancer patients reported similar patterns of sexual activity and responsiveness as the healthy sample. With the appearance of disease signs, however, the gynecologic cancer patients reported experiencing significant sexual dysfunction symptoms. While sexual morbidity is typically conceptualized as occurring after the diagnosis and treatment of cancer, these data indicate that such changes are a major source of variation in describing the prediagnosis sexual status of the gynecologic cancer patient.

  url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2705956/

Record 1694

pmcid: PMC2328236

pmid: 21267320

doi:

title: Sexuality during and after Pregnancy.

authors

- Still H

  journal: Canadian family physician Medecin de famille canadien

  journalAbbr: Can Fam Physician

  year: 1986

  pubdate: 1986 Oct

  volume: 32

  issue:

  pages: 2177-9

  abstract: Most standard obstetrical and gynecological textbooks ignore the subject of sexuality during and after pregnancy, and physicians who practise obstetrics tend to pay scant attention to this aspect of their patients' well-being. Since 1966, when Masters and Johnson's studies were published, relatively little information about pregnancy and sexuality has appeared in the medical literature. A comprehensive survey of the feelings and opinions of some 300 couples, published in 1977, indicates patients' desire and need to discuss sexual concerns with their doctor during and after pregnancy. This article summarizes the relevant literature and discusses the author's own experience of counselling his maternity patients. It also addresses problems which may develop in the marital relationship when a baby enters the pre-existing marital dyad.

  url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2328236/

Record 1695

pmcid: PMC1491277

pmid: 3756730

doi:

title: Student interest in family medicine careers has dropped drastically.

authors

- Sullivan P

journal: CMAJ : Canadian Medical Association journal = journal de l'Association medicale canadienne

journalAbbr: CMAJ

year: 1986

pubdate: 1986 Nov 1

volume: 135

issue: 9

pages: 1014-5

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1491277/

Record 1696

pmcid: PMC2571430

pmid: 3491906

doi:

title: Suicide and black adolescents: a medical dilemma.

authors

- Smith JA 3rd
- Carter JH

journal: Journal of the National Medical Association

journalAbbr: J Natl Med Assoc

year: 1986

pubdate: 1986 Nov

volume: 78

issue: 11

pages: 1061-4

abstract: The psychosocial dynamics of black adolescent suicide are examined in light of the available literature and the authors' clinical experience. The authors contend that members of the medical profession who are in touch with the physical problems of black adolescents must become sensitive to the magnitude of the problem and assist with developing compassionate, bold, and responsible suicide prevention and treatment modalities.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2571430/

Record 1697

pmcid: PMC1477645

pmid: 3080800

doi:

title: Testicular self-examination: a public awareness program.

authors

- Carlin PJ

journal: Public health reports (Washington, D.C. : 1974)

journalAbbr: Public Health Rep

year: 1986

pubdate: 1986 Jan-Feb

volume: 101

issue: 1

pages: 98-102

abstract: Testicular cancer is responsible for one out of seven deaths among late adolescent and young adult males. It is the number one cancer killer of men in the second and third decade of life. However, the population at risk, men between the ages 15 and 35, is singularly unaware of this cancer's existence and its potential danger. A public awareness program has been designed to increase consciousness about this possibly fatal disease and to enhance early detection and cure through a program of testicular self-examination (TSE). This proposal incorporates intervention at local, State, and national levels. It contemplates an innovative campaign beginning at the local level, focusing on the target population and utilizing a collaboration between nonprofessionals and professionals. The aim is for a grass-roots beginning and continued public interest to culminate in a national campaign, similar to that of breast self-examination. The ultimate goal of the program is to prevent the loss of lives to testicular cancer through public education and through use of TSE.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1477645/

Record 1698

pmcid: PMC2328239

pmid: 21267318

doi:

title: The Family Physician's Role in Preventing Early Termination of Breastfeeding.

authors

- Livingstone VH

journal: Canadian family physician Medecin de famille canadien

journalAbbr: Can Fam Physician

year: 1986

pubdate: 1986 Oct

volume: 32

issue:

pages: 2162-9

abstract: Despite the high rate of breastfeeding among mothers as they leave the hospital, early termination of breastfeeding continues to be a problem. A new mother needs considerable education, support and, often, early intervention, not only to initiate breastfeeding successfully, but also to prevent breastfeeding problems occurring and to continue breastfeeding successfully for several months. Knowledgeable health-care providers are the key to promoting and protecting breastfeeding, yet in the community, many of these mothers, lacking the support of such knowledgeable advisers, often terminate early. Family physicians are in a key position to help the nursing dyad. This article looks at the role that family physicians can play in advising and helping mothers, and discusses the management of common breastfeeding difficulties.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2328239/

Record 1699

pmcid: PMC1491535

pmid: 3091235

doi:

title: US groups eyeing merits of Canadian-style national health program.

authors

- Korcok M

journal: CMAJ : Canadian Medical Association journal = journal de l'Association medicale canadienne

journalAbbr: CMAJ

year: 1986

pubdate: 1986 Sep 1

volume: 135

issue: 5

pages: 536-41

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1491535/

Record 1700

pmcid: PMC425530

pmid: 3924954

doi: 10.1172/JCI111888

title: Familial gynecomastia with increased extraglandular aromatization of plasma carbon19-steroids.

authors

- Berkovitz GD
- Guerami A
- Brown TR
- MacDonald PC
- Migeon CJ

journal: The Journal of clinical investigation

journalAbbr: J Clin Invest

year: 1985

pubdate: 1985 Jun

volume: 75

issue: 6

pages: 1763-9

abstract: We evaluated a family in which gynecomastia occurred in five males in two generations. In each affected subject, gynecomastia and male sexual maturation began at an early age. The ratio of the concentration of plasma estradiol-17 beta to that of plasma testosterone was elevated in each affected subject. In the three siblings with gynecomastia, the transfer constant of conversion of androstenedione to estrone (i.e., the fraction of plasma androstenedione that was converted to estrone as measured in the urine) was 10 times that of normal persons. The transfer constant of conversion of testosterone to estradiol-17 beta in the one

subject studied also was 8-10 times that of normal men, whereas the transfer constants of conversion of estrone to estradiol-17 beta and of estradiol-17 beta to estrone were normal. Despite the elevation in extraglandular aromatase activity, there was a normal response of the hypothalamic-pituitary axis to provocative stimuli. This is the second documentation of gynecomastia that is associated with increased extraglandular aromatase activity, and the first time that the defect was found to be familial with a probable X-linked (or autosomal dominant, sex limited) mode of inheritance.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC425530/

Record 1701

pmcid: PMC2418276

pmid: 4022882

doi: 10.1136/pgmj.61.715.441

title: Female type cystic hyperplasia in a male breast.

authors

- McClure J
- Banerjee SS
- Sandilands DG

journal: Postgraduate medical journal

journalAbbr: Postgrad Med J

year: 1985

pubdate: 1985 May

volume: 61

issue: 715

pages: 441-3

abstract: A 28 year old man had a swelling of the right breast. He was phenotypically and karyotypically a normal male. Microscopic examination of the excised specimen showed the pattern of cystic hyperplasia identical to that commonly seen in the female breast. Numerous cysts were grouped in a lobular pattern and their lining showed apocrine metaplasia. There was extensive background fibrosis, some mammary duct ectasia and some features consistent with the more classical microscopic appearances of gynaecomastia. Whilst some of the features noted above are sometimes seen in gynaecomastia the presence of the full range of the features of cystic hyperplasia is distinctly unusual in the male.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2418276/

Record 1702

pmcid: PMC1345876

pmid:

doi:

title: Newsbriefs.

authors

journal: Canadian Medical Association Journal

journalAbbr: Can Med Assoc J

year: 1985

pubdate: 1985 Apr 1

volume: 132

issue: 7

pages: 825-32

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1345876/

Record 1703

pmcid: PMC1418278

pmid: 3919879

doi: 10.1136/bmj.290.6473.984

title: Paediatrics among ethnic minorities. Afro-Caribbean and African families.

authors

- Black J

journal: British medical journal (Clinical research ed.)

journalAbbr: Br Med J (Clin Res Ed)

year: 1985

pubdate: 1985 Mar 30

volume: 290

issue: 6473

pages: 984-8

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1418278/

Record 1704

pmcid: PMC1777679

pmid: 4091585

doi: 10.1136/adc.60.12.1180

title: Premature thelarche: a follow up study of 40 girls. Natural history and endocrine findings.

authors

- Pasquino AM
- Tebaldi L
- Cioschi L
- Cives C
- Finocchi G
- Maciocci M
- Mancuso G
- Boscherini B

journal: Archives of disease in childhood

journalAbbr: Arch Dis Child

year: 1985

pubdate: 1985 Dec

volume: 60

issue: 12

pages: 1180-2

abstract: Follow up of 40 girls with premature thelarche showed that where this disorder occurred before age 2 years it usually regressed completely, thus representing a transient and isolated phenomenon. Premature thelarche after age 2 years persisted more frequently, however, and represented the first sign of sexual development, generally leading to simple early puberty.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1777679/

Record 1705

pmcid: PMC1959977

pmid: 3989779

doi:

title: Puberty gynaecomastia.

authors

- Hooper PD

journal: The Journal of the Royal College of General Practitioners

journalAbbr: J R Coll Gen Pract

year: 1985

pubdate: 1985 Mar

volume: 35

issue: 272

pages: 142

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1959977/

Record 1706

pmcid: PMC1250946

pmid: 3901941

doi: 10.1097/00000658-198510000-00007

title: The surgical treatment of fibrous dysplasia. With emphasis on recent contributions from cranio-maxillo-facial surgery.

authors

- Edgerton MT
- Persing JA
- Jane JA

journal: Annals of surgery

journalAbbr: Ann Surg

year: 1985

pubdate: 1985 Oct

volume: 202

issue: 4

pages: 459-79

abstract: Fibrous dysplasia is a congenital, metabolic, nonfamilial disturbance that occurs in one or more bones, at times in association with skin pigmentations or endocrine abnormalities. The authors report on a large personal series of 23 patients with fibrous dysplasia involving the craniofacial skeleton. The etiology, clinical findings, pathology, and differential diagnosis of this condition are reviewed and a working hypothesis is offered for the pathophysiology of this disorder. Approximately one-third of patients with fibrous dysplasia have involvement of the cranial or facial bones. The authors describe how new techniques in craniofacial surgery have opened up additional options for this group of patients. Deformity, diplopia, proptosis, sinus infection, deafness, and loss of vision, are some of the clinical features that may require early surgical management. Evidence is given to support more complete resection of bony lesions with immediate reconstruction by several techniques. The removal, remodeling, and replacement of the dysplastic bone is advanced as a promising new method for the management of these complex problems. Successful use of this technique in four patients is reported. In a separate group of patients, continuing good experience is reported with cranio-orbital reconstruction by means of large methyl-methacrylate implants. Both of these surgical approaches eliminate donor site morbidity that results from the grafting of large amounts of autogenous bone. Both techniques also avoid the problems associated with postoperative absorption of bone grafting. Several patients are reported in whom serious disturbances in visual function appear to have been prevented or reversed by early treatment. Factors leading to malignant change in patients with fibrous dysplasia are reviewed.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1250946/

Record 1707

pmcid: PMC1777450

pmid: 3901933

doi: 10.1136/adc.60.9.819

title: Transient sexual precocity and ovarian cysts.

authors

- Lyon AJ
- De Bruyn R
- Grant DB

journal: Archives of disease in childhood

journalAbbr: Arch Dis Child

year: 1985

pubdate: 1985 Sep

volume: 60

issue: 9

pages: 819-22

abstract: Nine girls presenting under the age of 7 years with unsustained sexual precocity are described. Large ovarian cysts were detected by ultrasound in three and laparotomy in one. In two girls the symptoms resolved after surgical removal of the cyst; the other seven had spontaneous remission of symptoms, but in two of these transient breast development and bleeding recurred: further ovarian cyst formation was found in one of these patients. Endocrine studies performed before resolution of the cysts showed raised plasma oestradiol concentrations (64 to 440 pmol/l) in three girls and no appreciable rise in plasma luteinising hormone after gonadotrophin releasing hormone stimulation in two. We conclude that ovarian cyst formation with spontaneous resolution may cause transient sexual precocity in girls, and that ultrasound examination is an effective means of diagnosing and following these patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1777450/

Record 1708

pmcid: PMC1424718

pmid: 3918328

doi:

title: Women's health. Report of the Public Health Service Task Force on Women's Health Issues.

authors

journal: Public health reports (Washington, D.C. : 1974)

journalAbbr: Public Health Rep

year: 1985

pubdate: 1985 Jan-Feb

volume: 100

issue: 1

pages: 73-106

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1424718/

Record 1709

pmcid: PMC425420

pmid: 6542571

doi: 10.1172/JCI111654

title: Antiestrogenic action of dihydrotestosterone in mouse breast. Competition with estradiol for binding to the estrogen receptor.

authors

- Casey RW
- Wilson JD

journal: The Journal of clinical investigation

journalAbbr: J Clin Invest

year: 1984

pubdate: 1984 Dec

volume: 74

issue: 6

pages: 2272-8

abstract: Feminization in men occurs when the effective ratio of androgen to estrogen is lowered. Since sufficient estrogen is produced in normal men to induce breast enlargement in the absence of adequate amounts of circulating androgens, it has been generally assumed that androgens exert an antiestrogenic action to prevent feminization in normal men. We examined the mechanisms of this effect of androgens in the mouse breast. Administration of estradiol via silastic implants to castrated virgin CBA/J female mice results in a doubling in dry weight and DNA content of the breast. The effect of estradiol can be inhibited by implantation of 17 beta-hydroxy-5 alpha-androstan-3-one (dihydrotestosterone), whereas dihydrotestosterone alone had no effect on breast growth. Estradiol administration also enhances the level of progesterone receptor in mouse breast. Within 4 d of castration, the progesterone receptor virtually disappears and estradiol treatment causes a twofold increase above the level in intact animals. Dihydrotestosterone does not compete for binding to the progesterone receptor, but it does inhibit estrogen-mediated increases of progesterone receptor content of

breast tissue cytosol from both control mice and mice with X-linked testicular feminization (tfm)/Y. Since tfm/Y mice lack a functional androgen receptor, we conclude that this antiestrogenic action of androgen is not mediated by the androgen receptor. Dihydrotestosterone competes with estradiol for binding to the cytosolic estrogen receptor of mouse breast, whereas 17 beta-hydroxy-5 beta-androstan-3-one (5 beta-dihydrotestosterone) neither competes for binding nor inhibits estradiol-mediated induction of the progesterone receptor. Dihydrotestosterone also promotes the translocation of estrogen receptor from cytoplasm to nucleus; the ratio of cytoplasmic-to-nuclear receptor changes from 3:1 in the castrate to 1:2 in dihydrotestosterone-treated mice. Thus, the antiestrogenic effect of androgen in mouse breast may be the result of effects of dihydrotestosterone on the estrogen receptor. If so, dihydrotestosterone performs one of its major actions independent of the androgen receptor.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC425420/

Record 1710

pmcid: PMC2418046

pmid: 6494084

doi: 10.1136/pgmj.60.708.653

title: Breast problems in 1,000 consecutive referrals to surgical out-patients.

authors

- Ellis H
- Cox PJ

journal: Postgraduate medical journal

journalAbbr: Postgrad Med J

year: 1984

pubdate: 1984 Oct

volume: 60

issue: 708

pages: 653-6

abstract: One-thousand consecutive referrals of breast complaints to surgical out-patients are presented with breakdown according to the pathological groups of final diagnoses. The relationship of these groups to parity, breast feeding and age are discussed as also is the clinical management of these cases.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2418046/

Record 1711

pmcid: PMC1424620

pmid: 6435165

doi:

title: Epidemic of gynecomastia among illegal Haitian entrants.

authors

- Sattin RW
- Roisin A
- Kafrissen ME
- Dugan JB
- Farer LS

journal: Public health reports (Washington, D.C. : 1974)

journalAbbr: Public Health Rep

year: 1984

pubdate: 1984 Sep-Oct

volume: 99

issue: 5

pages: 504-10

abstract: Gynecomastia may occur as a normal physiologic development at certain ages or as a result of a variety of pathological conditions. An outbreak of gynecomastia was investigated at two processing centers of the Immigration and Naturalization Service (INS) between December 2, 1981, and May 14, 1982. At the Fort Allen Service Processing Center, Puerto Rico, gynecomastia was initially detected in 77 of 540 Haitian male entrants (14 percent) and in only 6 of 186 male employees of the center (3 percent) who were 18-50 years old; the difference in prevalence was statistically significant. At the Krome North Service Processing Center in Miami, Fla., gynecomastia was initially detected in 52 of 512 Haitian males 18-50 years old (10 percent). Two case-control studies did not demonstrate an association between gynecomastia and a number of factors that might have been related to an exogenous estrogen or to a substance with an estrogenic effect. Estrogen or estrogen-like substances were not found in food, water, or environmental samples. When the populations were rescreened several months later, 76 of the persons with gynecomastia detected in the first screening had had total or partial remission. Persons with remission had arrived earlier--a mean of 21.6 days for those at Fort Allen and 36.7 for those at Krome--than did those with newly detected gynecomastia and those with continuing cases. The difference in arrival dates was significant (P less than .005 for Fort Allen and P less than .001 for Krome). These results, in view of nutritional deprivation in Haiti, suggest that these cases may have been an outbreak of refeeding gynecomastia.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1424620/

Record 1712

pmcid: PMC1049238

pmid: 6716410

doi: 10.1136/jmg.21.2.114

title: Familial occurrence of unilateral giant breasts in Nigeria: a possible new genetic entity.

authors

- Badejo OA

journal: Journal of medical genetics

journalAbbr: J Med Genet

year: 1984

pubdate: 1984 Apr

volume: 21

issue: 2

pages: 114-6

abstract: Four cases of unilateral giant breasts from two unrelated families are described. Three of the patients were managed surgically. It is speculated from a review of available published reports that this condition may be genetic with an autosomal dominant mode of inheritance.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1049238/

Record 1713

pmcid: PMC1353486

pmid: 6721603

doi: 10.1097/00000658-198405000-00008

title: Hyperadrenalism in childhood and adolescence.

authors

- Thomas CG Jr
- Smith AT
- Griffith JM
- Askin FB

journal: Annals of surgery

journalAbbr: Ann Surg

year: 1984

pubdate: 1984 May

volume: 199

issue: 5

pages: 538-48

abstract: Hyperadrenalism in childhood and adolescence has unique features that influence diagnosis and management. We reviewed our experience with 18 patients, ranging in age from 18 months to 18 years. Nine had bilateral adrenal hyperplasia, eight had adrenal neoplasms, and one had micronodular hyperplasia. Patients with congenital adrenal hyperplasia and hyperaldosteronism were excluded. Six patients with Cushing's disease diagnosed in earlier years were treated by total adrenalectomy and recently two patients underwent transsphenoidal removal of pituitary tumors. Bilateral adrenalectomy was carried out in one patient with micronodular hyperplasia and in a second because of elevated adrenocorticotrophic hormone (ACTH) levels from an undefined source. Eight patients had adrenal neoplasms, including five adenomas and three carcinomas. We found no reliable criteria to differentiate before surgery between adrenal adenomas and adrenal carcinomas. The most recognizable characteristic of malignancy was tumor size, specifically weight greater than 75 gms. Of the three patients with adrenal carcinoma, one expired 20 months after adrenalectomy and 8 months after receiving palliative partial hepatectomy for liver metastasis. Two patients are well with normal growth and development at 11 and 20 years following adrenalectomy. With the exception of one patient who died 6 years after surgery from a glioblastoma multiforme, all patients with adrenal adenomas are well. Eight patients underwent bilateral adrenalectomy for hypercortisolism. Five of the six who have reached their adult stature are significantly stunted. Four of six patients with Cushing's disease, treated by total adrenalectomy, have developed Nelson's syndrome at 2, 6, 10, and 12 years after surgery. Of the two patients undergoing transsphenoidal surgery, one had recurrent disease at 2 years and was treated by pituitary irradiation with recovery. The patients undergoing adrenalectomy for micronodular hyperplasia and ectopic ACTH are well at 2 and 4 years, respectively. Cushing's disease in children and adolescents is best treated by transsphenoidal removal of the pituitary adenoma. Adrenalectomy, once the most accepted approach, plays a secondary role and is indicated primarily in micronodular adrenal hyperplasia, in patients with ectopic ACTH production of an undefined source, and in recurrent Cushing's disease following prior pituitary irradiation. The high incidence of Nelson's syndrome in children treated by adrenalectomy mandates that patients at risk be monitored lifelong for the progression of a pituitary tumor.(ABSTRACT TRUNCATED AT 400 WORDS)

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1353486/

Record 1714

pmcid: PMC1442021

pmid: 6430394

doi: 10.1136/bmj.289.6438.160-a

title: Spontaneous separation of fused labia minora in prepubertal girls.

authors

- Jenkinson SD
- MacKinnon AE

journal: British medical journal (Clinical research ed.)

journalAbbr: Br Med J (Clin Res Ed)

year: 1984

pubdate: 1984 Jul 21

volume: 289

issue: 6438

pages: 160-1

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1442021/

Record 1715

pmcid: PMC2153656

pmid:

doi:

title: Administrative content — journal masthead, notices, indexes, etc.

authors

journal: Can Fam Physician

journalAbbr: Can Fam Physician

year: 1983

pubdate: 1983 Jun

volume: 29

issue:

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2153656/

Record 1716

pmcid: PMC2589666

pmid: 6659560

doi:

title: Diabetes and the stomach.

authors

- Saltzman MB
- McCallum RW

journal: The Yale journal of biology and medicine

journalAbbr: Yale J Biol Med

year: 1983

pubdate: 1983 May-Jun

volume: 56

issue: 3

pages: 179-87

abstract: Abnormalities in the function of the stomach in patients with long-standing diabetes mellitus, usually insulin-dependent, may provide difficult management problems. There is a reduced frequency of peptic ulcer disease in diabetics. Gastric atrophy, often with parietal cell antibodies, is common and the frequency of pernicious anemia with its expected intrinsic factor antibodies is increased. Gastric analysis results have been conflicting but generally suggest that long-standing diabetics have lower acid levels than normals, possibly secondary to vagal neuropathy. Gastric atony occurring in a small but significant number of patients with longstanding insulin-dependent diabetes, usually with a clinically apparent peripheral neuropathy, has been associated with upper abdominal discomfort, vomiting, and a clinical picture of gastric outlet obstruction. Various degrees of subclinical delays in gastric emptying are probably present in many asymptomatic patients and, indeed, are underemphasized contributors to poor control of blood sugar levels. Studies utilizing radioactive-labeled physiological meals have demonstrated abnormalities in the gastric emptying of solids, in particular, and sometimes liquids in the latter stages of the disease. Metoclopramide, a dopamine antagonist, which stimulates upper gastrointestinal smooth musculature, results in accelerated gastric emptying; clinical trials have shown that it is capable of alleviating symptoms related to diabetic gastroparesis and with its recent approval and release in this country, it promises improved management of this entity. Another agent, domperidone, a selective peripheral dopamine antagonist with no appreciable side effects, is in this country an investigational drug which has shown clinical efficacy in Europe in improving gastric stasis syndromes.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2589666/

Record 1717

pmcid: PMC1010808

pmid: 6880188

doi:

title: Prolactinoma in a man following industrial exposure to estrogens.

authors

- Baron SH
- Sowers JR
- Feinberg M

journal: The Western journal of medicine

journalAbbr: West J Med

year: 1983

pubdate: 1983 May

volume: 138

issue: 5

pages: 720-2

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1010808/

Record 1718

pmcid: PMC1498323

pmid: 6282377

doi: 10.1136/bmj.284.6327.1440

title: Adverse reactions to nitrofurantoin in the United Kingdom, Sweden, and Holland.

authors

- Penn RG
- Griffin JP

journal: British medical journal (Clinical research ed.)

journalAbbr: Br Med J (Clin Res Ed)

year: 1982

pubdate: 1982 May 15

volume: 284

issue: 6327

pages: 1440-2

abstract: A study was carried out comparing the reporting of adverse reactions to nitrofurantoin in Sweden, Holland, and the United Kingdom. The Swedish drug regulatory authorities receive more reports of adverse reactions to nitrofurantoin than to any other drug, and the annual reporting rate is increasing despite a steady fall in the prescription of nitrofurantoin. In the United Kingdom the total number of adverse reaction to nitrofurantoin reported to the Committee on Safety of Medicines has been steadily falling for many years, as has the number of prescription. Lung reactions and liver damage are more commonly reported in Sweden than in the UK, where gastrointestinal symptoms and peripheral neuropathy are more common. Such figures as were available in Holland showed an intermediate picture. The incidences of blood dyscrasias and allergic reactions are similar in the three countries. The causes for such differences are unresolved, but it is disturbing that the incidence and range of adverse reactions to such a common drug as nitrofurantoin reported to the national authorities can differ so much in countries that have close ethnic, dietary, and economic similarities. Thus great caution must be observed when data on adverse drug reactions from the country are extrapolated to another.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1498323/

Record 1719

pmcid: PMC1500445

pmid: 6814578

doi: 10.1136/bmj.285.6352.1405

title: Essentials of health economics: part IV--Organising health care resources.

authors

- Mooney GH
- Drummond MF

journal: British medical journal (Clinical research ed.)

journalAbbr: Br Med J (Clin Res Ed)

year: 1982

pubdate: 1982 Nov 13

volume: 285

issue: 6352

pages: 1405-6

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1500445/

Record 1720

pmcid: PMC2306417

pmid: 21286106

doi:

title: Exercise and the asymptomatic individual: assessment and advice.

authors

- Skrastins R
- McCans JL

journal: Canadian family physician Medecin de famille canadien

journalAbbr: Can Fam Physician

year: 1982

pubdate: 1982 May

volume: 28

issue:

pages: 974-9

abstract: With the current popularity of physical fitness, the family physician is often asked to advise asymptomatic individuals who wish to undertake an exercise program. In the majority of cases, adequate assessment consists of a thorough history and physical examination, along with a few simple investigations, including a resting electrocardiogram. Exercise stress testing of asymptomatic individuals produces an unacceptably high frequency of false-positive results, and its use should be restricted to those patients with cardiac symptoms or major cardiac risk factors. The potential benefits of a longterm commitment to regular exercise should be discussed with the patient and guidance provided on the optimal form of exercise program for that individual. Exercise must not be considered in isolation. Other major cardiovascular risk factors should be sought and dealt with appropriately.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2306417/

Record 1721

pmcid: PMC1500634

pmid:

doi:

title: INDEX TO VOLUME 285 FOR 1982.

authors

journal: British Medical Journal (Clinical research ed.)

journalAbbr: Br Med J (Clin Res Ed)

year: 1982

pubdate: 1982 Dec

volume: 285

issue: Suppl

pages: 3-39

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1500634/

Record 1722

pmcid: PMC1496298

pmid: 6802330

doi: 10.1136/bmj.284.6319.868

title: Oral contraception in patients with hyperprolactinaemia.

authors

- Moult PJ
- Dacie JE
- Rees LH
- Besser GM

journal: British medical journal (Clinical research ed.)

journalAbbr: Br Med J (Clin Res Ed)

year: 1982

pubdate: 1982 Mar 20

volume: 284

issue: 6319

pages: 868

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1496298/

Record 1723

pmcid: PMC1627596

pmid: 6462113

doi: 10.1136/adc.57.3.200

title: Premature thelarche: a possible adrenal disorder.

authors

- Dumic M
- Tajic M
- Mardesic D
- Kalafatic Z

journal: Archives of disease in childhood

journalAbbr: Arch Dis Child

year: 1982

pubdate: 1982 Mar

volume: 57

issue: 3

pages: 200-3

abstract: Endocrine studies in girls with precocious thelarche were compared with those of normal girls of similar ages. Girls with precocious thelarche showed breast development and oestrogenised vaginal smears as the only signs of precocious sexual development. A few of the girls were tall and some had advanced bone ages but these two findings were not consistently present in the same patient. Hormones--such as serum oestradiol,

oestrone, delta 4-androstenedione, progesterone, dehydroepiandrosterone (DHEA), follicle-stimulating hormone, luteinising hormone, and prolactin, and urinary 17-ketosteroids--were measured. Only DHEA was different, being higher in girls with precocious thelarche. It is suggested that the high DHEA level may serve as a precursor for conversion to oestrogens in target tissues, breast, and vagina. This mechanism for oestrogenisation had been reported in other patients.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1627596/

Record 1724

pmcid: PMC1500056

pmid: 6812755

doi: 10.1136/bmj.285.6348.1083-a

title: Reversible gynaecomastia associated with D-penicillamine in a man with rheumatoid arthritis.

authors

- Reid DM
- Martynoga AG
- Nuki G

journal: British medical journal (Clinical research ed.)

journalAbbr: Br Med J (Clin Res Ed)

year: 1982

pubdate: 1982 Oct 16

volume: 285

issue: 6348

pages: 1083-4

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1500056/

Record 1725

pmcid: PMC1628083

pmid: 6758707

doi: 10.1136/adc.57.12.954

title: Transient pseudo-precocious puberty by probable oestrogen intake in 3 girls.

authors

- Pasquino AM
- Balducci R
- Manca Bitti ML
- Spadoni GL
- Boscherini B

journal: Archives of disease in childhood

journalAbbr: Arch Dis Child

year: 1982

pubdate: 1982 Dec

volume: 57

issue: 12

pages: 954-6

abstract: We report the clinical and laboratory findings in 3 prepubertal girls with transient signs of sexual precocity. Accidental oestrogen intake from contaminated food was the most likely cause, the luteinsing hormone—releasing hormone test showing suppressed secretion of follicle-stimulating hormone and luteinsing hormone at the time of maximal oestrogenisation.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1628083/

Record 1726

pmcid: PMC2306209

pmid:

doi:

title: CMA response to Task Force report.

authors

journal: Canadian Family Physician

journalAbbr: Can Fam Physician

year: 1981

pubdate: 1981 Oct

volume: 27

issue:

pages: 1513-7

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2306209/

Record 1727

pmcid: PMC491211

pmid: 7334392

doi: 10.1136/jnnp.44.11.1046

title: Further observations on the "bobble-headed doll syndrome".

authors

- Dell S

journal: Journal of neurology, neurosurgery, and psychiatry

journalAbbr: J Neurol Neurosurg Psychiatry

year: 1981

pubdate: 1981 Nov

volume: 44

issue: 11

pages: 1046-9

abstract: Two patients are presented with a head-bobblng "tic" classically associated with a suprasellar cyst. The first of these showed hypothalamic, neuroendocrinological and limbic abnormalities not previously described, although with a classic arachnoidal suprasellar cyst. The second of these patients showed head-bobbling behaviour during the first years of life as a manifestation of one out of many episodes of ventricular

shunt obstruction. This case is therefore the first reported whose head bobbling was observed with an acute lesion.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC491211/

Record 1728

pmcid: PMC181611

pmid: 6267997

doi: 10.1128/AAC.19.6.1073

title: Gynecomastia with ketoconazole.

authors

- DeFelice R
- Johnson DG
- Galgiani JN

journal: Antimicrobial agents and chemotherapy

journalAbbr: Antimicrob Agents Chemother

year: 1981

pubdate: 1981 Jun

volume: 19

issue: 6

pages: 1073-4

abstract: Three of forty men developed bilateral gynecomastia upon ketoconazole treatment. Symptoms abated despite continued therapy. This appears to be a direct drug effect on breast tissue.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC181611/

Record 1729

pmcid: PMC1627313

pmid: 7305415

doi: 10.1136/adc.56.10.770

title: Prolactin, maternal oestrogens, and breast development in the newborn.

authors

- McKiernan JF
- Hull D

journal: Archives of disease in childhood

journalAbbr: Arch Dis Child

year: 1981

pubdate: 1981 Oct

volume: 56

issue: 10

pages: 770-4

abstract: To investigate the influence of maternal oestrogens on the fetal breast development, maternal urinary oestriol excretion, maternal plasma oestriol concentrations, and cord venous plasma oestradiol and oestriol

concentrations were related to the size of the neonatal breast. A significant positive association between oestriol excretion and neonatal breast size was demonstrated, but the relationship was not strong and might be due to both measures having a positive relationship with birthweight. The infants' circulating concentrations of prolactin at birth and during the first weeks of life were also related to breast size. There was no cord venous-arterial difference in prolactin concentrations, and neither related to breast diameter. However there was a strong association between breast size and prolactin concentrations in mature infants aged between 5 and 7 days. In preterm infants breast tissue often develops after birth. Prolactin levels in preterm infants were higher between 2 and 6 weeks than they were in the first week of life. It would appear that the early development of the breast is influenced more by the infants's than the mother's endocrine activity.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1627313/

Record 1730

pmcid: PMC1504185

pmid: 6780026

doi: 10.1136/bmj.282.6261.362-a

title: Successful treatment of D-penicillamine-induced breast gigantism with danazol.

authors

- Taylor PJ
- Cumming DC
- Corenblum B

journal: British medical journal (Clinical research ed.)

journalAbbr: Br Med J (Clin Res Ed)

year: 1981

pubdate: 1981 Jan 31

volume: 282

issue: 6261

pages: 362-3

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1504185/

Record 1731

pmcid: PMC1505498

pmid: 6786588

doi: 10.1136/bmj.282.6276.1627-e

title: Successful treatment of D-penicillamine-induced breast gigantism with danazol.

authors

- Rooney PJ
- Cleland J

journal: British medical journal (Clinical research ed.)

journalAbbr: Br Med J (Clin Res Ed)

year: 1981

pubdate: 1981 May 16

volume: 282

issue: 6276

pages: 1627-8

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1505498/

Record 1732

pmcid: PMC1000489

pmid:

doi:

title: Administrative content — journal masthead, notices, indexes, etc.

authors

journal: Ann Rheum Dis

journalAbbr: Ann Rheum Dis

year: 1980

pubdate: 1980 Apr

volume: 39

issue: 2

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1000489/

Record 1733

pmcid: PMC1035082

pmid:

doi:

title: Administrative content — journal masthead, notices, indexes, etc.

authors

journal: Ann Rheum Dis

journalAbbr: Ann Rheum Dis

year: 1980

pubdate: 1980 Dec

volume: 39

issue: 6

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1035082/

Record 1734

pmcid: PMC1000511

pmid: 7189998

doi: 10.1136/ard.39.2.200-a

title: D penicillamine and breast enlargement.

authors

- Thew DC
- Stewart IM

journal: Annals of the rheumatic diseases

journalAbbr: Ann Rheum Dis

year: 1980

pubdate: 1980 Apr

volume: 39

issue: 2

pages: 200

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1000511/

Record 1735

pmcid: PMC2383830

pmid: 21293725

doi:

title: Failure to thrive: an approach to management.

authors

- Mawdsley JM
- Chaytors RG
- Higgins GL

journal: Canadian family physician Medecin de famille canadien

journalAbbr: Can Fam Physician

year: 1980

pubdate: 1980 Nov

volume: 26

issue:

pages: 1569-72

abstract: The family physician is often confronted with the problem of failure to thrive. The possible causes are numerous and diverse, so there is a tendency to attack the problem with a battery of expensive and sometimes unnecessary investigations, which serve only to upset the patient and contribute to the rising costs of health care. The use of a simple organized approach allows common diagnoses to be made with a minimum of costly investigations. In more complicated cases the use of this plan allows the family physician to pursue the investigation further, narrowing down the diagnostic possibilities before a consultation is required.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2383830/

Record 1736

pmcid: PMC1600492

pmid: 20793015

doi: 10.1136/bmj.280.6206.27

title: Letter from Chicago: Temples of doom.

authors

- Dunea G

journal: British medical journal

journalAbbr: Br Med J

year: 1980

pubdate: 1980 Jan 5

volume: 280

issue: 6206

pages: 27-8

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1600492/

Record 1737

pmcid: PMC1704998

pmid: 7448666

doi:

title: Perceived side effects of oral contraceptives among adolescent girls.

authors

- Herold ES
- Goodwin MS

journal: Canadian Medical Association journal

journalAbbr: Can Med Assoc J

year: 1980

pubdate: 1980 Nov 22

volume: 123

issue: 10

pages: 1022-6

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1704998/

Record 1738

pmcid: PMC1713724

pmid: 7407488

doi: 10.1136/bmj.281.6232.29

title: The prepregnancy clinic.

authors

- Chamberlain G

journal: British medical journal

journalAbbr: Br Med J

year: 1980

pubdate: 1980 Jul 5

volume: 281

issue: 6232

pages: 29-30

abstract: A prepregnancy clinic was set up to provide a single referral centre where women not yet pregnant but who were worried about problems when they became so could obtain advice. In the first 18 months 56 women attended. The outstanding feature is the concern of many women and their doctors about previous premature labour or late spontaneous abortion.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1713724/

Record 1739

pmcid: PMC1885918

pmid: 6937619

doi: 10.1136/jmg.17.6.437

title: X;Y translocation in an adolescent mentally normal phenotypic male with features of hypogonadism.

authors

- Bernstein R
- Pinto MR
- Almeida M
- Solarsh SM
- Meck J
- Jenkins T

journal: Journal of medical genetics

journalAbbr: J Med Genet

year: 1980

pubdate: 1980 Dec

volume: 17

issue: 6

pages: 437-43

abstract: Cytogenetic studies on a 17-year-old phenotypic male, with short stature and clinical and hormonal features of hypogonadism similar to those of an XX male, revealed an X;Y translocation, karyotype, 46,Xt(X;Y)(p22;?p11?q11). He was H-Y antigen positive. X inactivation studies showed inactivation of the abnormal X in the majority of cells (60 to 70%) and inactivation of the normal X in the remaining cells. Gene marker studies, including Xg blood grouping, showed no anomalous segregation. This patient is the second reported male showing a positively identified X;Y tanslocation with no detectable free Y chromosome and provides further indirect evidence for an X-Y interchange in the aetiology of XX male sex reversal.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1885918/

Record 1740

pmcid: PMC2537488

pmid: 501753

doi:

title: Benign breast lesions.

authors

- Ajao OG

journal: Journal of the National Medical Association

journalAbbr: J Natl Med Assoc

year: 1979

pubdate: 1979 Sep

volume: 71

issue: 9

pages: 867-8

abstract: A review of 52 unselected cases of benign breast masses operated on at the University College Hospital Ibadan, showed that fibroadenoma was the most common benign lesion found, followed by fibrocystic disease of the breast. Fibroadenoma is most common in the second and third decades of life, usually in nulliparous young women, whereas fibrocystic disease is most common in the fifth decade, usually in parous women. Benign breast lesions are more common in the left breast than the right, and the most common location is the upper outer quadrant.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2537488/

Record 1741

pmcid: PMC1818814

pmid: 367559

doi:

title: Dr. Christiaan Barnard: renowned surgeon, egoist but an old-fashioned family doctor at heart. Interview by Robert MacNeil.

authors

- Rowe DJ

journal: Canadian Medical Association journal

journalAbbr: Can Med Assoc J

year: 1979

pubdate: 1979 Jan 6

volume: 120

issue: 1

pages: 98-9

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1818814/

Record 1742

pmcid: PMC1419438

pmid: 428828

doi: 10.1136/gut.20.2.154

title: Gynaecomastia associated with cimetidine.

authors

- Spence RW
- Celestin LR

journal: Gut

journalAbbr: Gut

year: 1979

pubdate: 1979 Feb

volume: 20

issue: 2

pages: 154-7

abstract: Gynaecomastia has occurred unilaterally or bilaterally in five out of 25 male duodenal ulcer patients after more than four months treatment with cimetidine 1.6 g daily. All elected to continue treatment to 12 months and their breast enlargement regressed rapidly and disappeared after stopping treatment. During treatment all patients were found to have normal concentrations of plasma testosterone and oestradiol, and serum prolactin was normal in the two patients measured. Excision biopsy of the subareolar tissue in one patient revealed histology typical of the florid stage of gynaecomastia. Blockade of androgen-responsive receptors receptors in the target organ appears to be the most likely mechanism involved.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1419438/

Record 1743

pmcid: PMC1545255

pmid:

doi:

title: Manual of Emergency Pediatrics.

authors

- Illingworth CM

journal: Archives of Disease in Childhood

journalAbbr: Arch Dis Child

year: 1979

pubdate: 1979 Mar

volume: 54

issue: 3

pages: 246-7

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1545255/

Record 1744

pmcid: PMC1818905

pmid:

doi:

title: Obituaries.

authors

journal: Canadian Medical Association Journal

journalAbbr: Can Med Assoc J

year: 1979

pubdate: 1979 Feb 17

volume: 120

issue: 4

pages: 502-3

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1818905/

Record 1745

pmcid: PMC1818976

pmid: 427695

doi:

title: Outflow of Canadian physicians highest ever in 1978.

authors

- Korcok M

journal: Canadian Medical Association journal

journalAbbr: Can Med Assoc J

year: 1979

pubdate: 1979 Apr 7

volume: 120

issue: 7

pages: 878-82

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1818976/

Record 1746

pmcid: PMC1819015

pmid:

doi:

title: Personal File.

authors

journal: Canadian Medical Association Journal

journalAbbr: Can Med Assoc J

year: 1979

pubdate: 1979 Feb 3

volume: 120

issue: 3

pages: 383-4

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1819015/

Record 1747

pmcid: PMC1598514

pmid: 86374

doi: 10.1136/bmj.1.6167.866

title: Some experience in an area health authority child health clinic.

authors

- Illingworth RS

journal: British medical journal

journalAbbr: Br Med J

year: 1979

pubdate: 1979 Mar 31

volume: 1

issue: 6167

pages: 866-9

abstract: Three years' experience as a doctor taking two clinics a week in an area health authority child health clinic was reviewed. A wide range of clinical conditions was seen, including: problems associated with feeding in breast- and bottle-fed infants; minor developmental abnormalities (mental, behavioural, and physical); surgical and orthopaedic conditions requiring treatment; medical conditions, mainly respiratory and alimentary infections, skin conditions, and problems of over-treatment for minor ailments; and minor genetic abnormalities. Mothers asked for advice on a wide range of topics, risks and benefits of immunisation being the most common. The clinic doctor needs a wide experience in paediatrics to deal with such problems. It is suggested that all lecturers in child health and paediatric and senior registrars should take one clinic a week for six months, and all medical students should attend some clinics as part of their paediatric training. Health visitors have an important role in helping the clinic doctor, but their training should be more realistic and appropriate facilities should be provided to keep them up to date in their work.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1598514/

Record 1748

pmcid: PMC1271866

pmid: 394481

doi:

title: Symposium on Adolescent Gynecology and Endocrinology. Part I: Physiology of sexual maturation and primary amenorrhea. Primary amenorrhea.

authors

- Federman DD

journal: The Western journal of medicine

journalAbbr: West J Med

year: 1979

pubdate: 1979 Nov

volume: 131

issue: 5

pages: 411-6

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1271866/

Record 1749

pmcid: PMC1818557

pmid: 153779

doi:

title: [Hormonal dynamics during pregnancy: critical discussion and clinical interpretations].

authors

- Bélisle S

journal: Canadian Medical Association journal

journalAbbr: Can Med Assoc J

year: 1978

pubdate: 1978 Dec 9

volume: 119

issue: 11

pages: 1307-9

abstract: Although the clinical interpretation of its results varies greatly among perinatologists, the dehydroepiandrosterone sulfate (DHEA-S) loading test has become an integral part of fetal monitoring in many centres dealing with high-risk pregnancies. Currently four main metabolic responses to administration of DHEA-S to the mother are monitored to predict the functional state of the fetoplacental unit: the metabolic clearance of DHEA-S, the metabolic clearance of DHEA-S into estradiol, the conversion of DHEA-S into estradiol or esterol, and the increase in the plasma concentrations of DHEA and androstenedione after administration of DHEA-S. This article critically reviews each of these responses and its possible clinical interpretation, and assesses the clinical future of the DHEA-S loading test.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1818557/

Record 1750

pmcid: PMC1603010

pmid:

doi:

title: Breast lumps in adolescent girls.

authors

- McCracken D

journal: British Medical Journal

journalAbbr: Br Med J

year: 1978

pubdate: 1978 Feb 18

volume: 1

issue: 6110

pages: 443

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1603010/

Record 1751

pmcid: PMC1818180

pmid: 638910

doi:

title: Effect of hydrochlorothiazide on urine saturation with brushite, in vitro collagen calcification by urine, and urinary inhibitors of collagen calcification.

authors

- Pylypchuk G
- Ehrig U
- Wilson DR

journal: Canadian Medical Association journal

journalAbbr: Can Med Assoc J

year: 1978

pubdate: 1978 Apr 8

volume: 118

issue: 7

pages: 792-6

abstract: To clarify further the beneficial effect of thiazide diuretics on recurrent calcium nephrolithiasis, the effect of short-term hydrochlorothiazide therapy on urine saturation with brushite (CaHPO(4).2H(2)O), in vitro collagen calcification by urine, and urinary inhibitors of calcification was studied.In 22 patients with idiopathic calcium oxalate/phosphate stones the urine calcium excretion decreased, the urine magnesium excretion increased and the urine magnesium/calcium ratio increased significantly ($P < 0.001$) during hydrochlorothiazide therapy. Supersaturation of the urine with brushite, which was present in 19 of the 22 patients, was reduced significantly ($P < 0.001$) in all during thiazide therapy, and to the undersaturated range in 16. The ability of urine to calcify collagen in vitro also decreased significantly ($P < 0.001$) during thiazide therapy, a change that correlated significantly ($r = 0.4513$, $P < 0.05$) with the decrease in brushite saturation. The concentration of urinary inhibitors of calcification, as determined with an in vitro collagen calcification system, was decreased significantly ($P < 0.01$) by thiazide therapy.It was concluded that, in addition to decreasing urine calcium excretion and increasing urine magnesium excretion, thiazide diuretics decrease the urinary brushite saturation and thus may prevent spontaneous nucleation or crystal growth, or both, of calcium phosphate. The ability of thiazides to decrease collagen calcification in vitro suggests that they may also prevent crystal growth on a nidus of organic matrix. Thiazides do not appear to act by increasing the excretion of urinary inhibitors of calcification.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1818180/

Record 1752

pmcid: PMC1819074

pmid: 743666

doi:

title: Hepatitis and hepatitis B surface antigen and antibody in dentists.

authors

- Berris B
- Feinman SV
- Sinclair JC
- Wrobel D

journal: Canadian Medical Association journal

journalAbbr: Can Med Assoc J

year: 1978

pubdate: 1978 Nov 4

volume: 119

issue: 9

pages: 1040-3

abstract: Dentists were surveyed regarding a history of hepatitis and the presence in the blood of hepatitis B surface antigen (HBsAg) and antibody (anti-HBs) to determine whether they were at high risk of exposure to hepatitis B virus. Of 288 Canadian dentists 5.2% gave a history of hepatitis after graduation. This proportion is similar to that for 1462 Ontario dentists (6.3%) and that for 3162 accountants (5.1%) who had previously completed a mailed questionnaire. One dentist (0.3%) was HBsAg-positive and 42 (14.6%) were anti-HBs-positive. Of 210 healthy volunteer blood donors matched for age, sex and ethnic origin with the group of dentists none was HBsAg-positive and 2.9% significantly fewer (P less than 0.005), were anti-HBs-positive. Among Ontario blood donors 0.3% were HBsAg-positive and 3% were anti-HBs-positive. Thus, in Canada, dentists are not at increased risk of acquiring clinical hepatitis or becoming carriers, but they are more likely than other groups to have anti-HBs in the blood. Among dentists from outside Canada a higher proportion had a history of hepatitis (10.3%) and were HBsAg-positive (1.6%), but approximately the same proportion were anti-HBs-positive (15.9%).

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1819074/

Record 1753

pmcid: PMC1818139

pmid: 679133

doi:

title: Medidal research in Canada: a threatened national resource.

authors

- Wilson DL

journal: Canadian Medical Association journal

journalAbbr: Can Med Assoc J

year: 1978

pubdate: 1978 Aug 12

volume: 119

issue: 3

pages: 266-73

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1818139/

Record 1754

pmcid: PMC1818405

pmid: 657035

doi:

title: Normal blood pressure in offspring of persons with essential hypertension.

authors

- Kaplan BS
- Fox H
- Seidman E
- Drummond KN

journal: Canadian Medical Association journal

journalAbbr: Can Med Assoc J

year: 1978

pubdate: 1978 Jun 10

volume: 118

issue: 11

pages: 1415-7

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1818405/

Record 1755

pmcid: PMC1818734

pmid:

doi:

title: Obituaries.

authors

journal: Canadian Medical Association Journal

journalAbbr: Can Med Assoc J

year: 1978

pubdate: 1978 May 6

volume: 118

issue: 9

pages: 1164-7

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1818734/

Record 1756

pmcid: PMC1818306

pmid: 679098

doi:

title: Prinzmetal's angina, normal coronary arteries and pericarditis.

authors

- Lewis JR
- Kisilevsky R
- Armstrong PW

journal: Canadian Medical Association journal

journalAbbr: Can Med Assoc J

year: 1978

pubdate: 1978 Jul 8

volume: 119

issue: 1

pages: 36-9

abstract: A woman with Prinzmetal's variant angina had spontaneous attacks of myocardial ischemia characterized by severe chest pain, hypotension, inferior ST-segment elevation, transient complete heart block and selective right ventricular dysfunction. Despite initial improvement following intravenous administration of atropine and sublingual administration of nitroglycerin she died of cardiogenic shock. Autopsy showed normal coronary arteries and acute pericarditis, more pronounced over the right side of the heart. It is postulated that the pericardial inflammation elicited severe spasm of the subjacent right coronary artery.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1818306/

Record 1757

pmcid: PMC1817990

pmid: 343905

doi:

title: Role of the adrenal cortex and sodium in the pathogenesis of human hypertension.

authors

- Genest J
- Nowaczynski W
- Boucher R
- Kuchel O

journal: Canadian Medical Association journal

journalAbbr: Can Med Assoc J

year: 1978

pubdate: 1978 Mar 4

volume: 118

issue: 5

pages: 538-49

abstract: After 30 years of continuous research into the mechanisms of human hypertension, we summarize the results obtained by the members of the multidisciplinary research group on hypertension of the Clinical Research Institute of Montreal on the disturbances of minerlocorticoid activity in a rigorously selected group of patients with early, mild essential hypertension. We attempt to integrate these findings with those of many other groups working on other aspects of hypertensive cardiovascular diseases. On the assumption that the increased peripheral resistance responsible for hypertension results from an imbalance or a disturbance of the equilibrium between the sympathetic nervous system and norepinephrine on one hand, and the vascular tone, sensitivity and responsiveness of the arterial smooth muscle to norepinephrine and to angiotensin II on the other hand, three models that fit the experimental and clinical facts as known at present are described.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1817990/

Record 1758

pmcid: PMC1818483

pmid: 688149

doi:

title: Silo-filler's disease.

authors

- Fleetham JA
- Munt PW
- Tunnicliffe BW

journal: Canadian Medical Association journal

journalAbbr: Can Med Assoc J

year: 1978

pubdate: 1978 Sep 9

volume: 119

issue: 5

pages: 482-4

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1818483/

Record 1759

pmcid: PMC1818769

pmid: 709476

doi:

title: Usefulness of three additional electrocardiographic chest leads (V7, V8, and V9) in the diagnosis of acute myocardial infarction.

authors

- Melendez LJ
- Jones DT
- Salcedo JR

journal: Canadian Medical Association journal

journalAbbr: Can Med Assoc J

year: 1978

pubdate: 1978 Oct 7

volume: 119

issue: 7

pages: 745-8

abstract: Additional electrocardiocardiographic chest leads (V7, V8, and V9) were used in 117 persons consecutively admitted to a coronary care unit. Among the 46 (39%) with a proven acute myocardial infarction the electrocardiograms (ECGs) of 9 (20%) showed ST-segment elevation or abnormal Q-waves, or both, in the

three additional leads. In six of the nine, such changes were associated with signs of anterolateral or inferior wall infarction (in three each) on the standard 12-lead ECG, but in the other three (7% of the 46) electrocardiographic changes diagnostic of acute myocardial infarction were found only on the additional chest leads; the last three had characteristic changes in serum enzyme concentrations. This study showed that additional chest leads are helpful in detecting myocardial injury of necrosis in areas of the heart not properly reflected on the standard 12-lead ECG.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1818769/

Record 1760

pmcid: PMC1880430

pmid: 73406

doi:

title: Tumour antigens associated with primary mediastinal choriocarcinoma.

authors

- Forest JC
- Talbot J
- Pagé M
- Loiselle JM

journal: Canadian Medical Association journal

journalAbbr: Can Med Assoc J

year: 1977

pubdate: 1977 Dec 17

volume: 117

issue: 12

pages: 1386-8

abstract: Various biochemical markers of cancer were investigated in two men aged 26 and 45 years with primary mediastinal choriocarcinoma. The daily excretion of urinary human chorionic gonadotropin (HCG) and the serum concentration of the beta-subunit of HCG were elevated in both patients, but carcinoembryonic antigen, Regan isoenzyme, alpha1-fetoprotein, serum pregnancy-associated globulin and human chorionic somatomammotropin were not detectable. Comparison of the results of the investigation of biochemical markers of this rare neoplasm in these two men with those published previously illustrates the discordance in the expression of biochemical markers of primary mediastinal choriocarcinoma.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1880430/

Record 1761

pmcid: PMC1639855

pmid: 773500

doi: 10.1136/bmj.1.6017.1052

title: Azathioprine and penicillamine in treatment of rheumatoid arthritis: a controlled trial.

authors

- Berry H
- Liyanage SP
- Durance RA
- Barnes CG

- Berger LA
- Evans S

journal: British medical journal

journalAbbr: Br Med J

year: 1976

pubdate: 1976 May 1

volume: 1

issue: 6017

pages: 1052-4

abstract: Sixty-five patients, 33 receiving azathioprine and 32 receiving penicillamine, took part in a one-year, single-blind external-observer trial designed to compare the efficacy and toxicity of these two drugs in the treatment of rheumatoid arthritis. By six months there was a significantly greater rise in haemoglobin and fall in erythrocyte sedimentation rate among those receiving penicillamine, and by one year this difference remained only in the increase in haemoglobin levels. Fifteen patients, 10 on azathioprine and 5 on penicillamine, had to stop treatment because of side effects; 90 single side effects occurred, 48 in those on penicillamine and 42 in those on azathioprine. After one year both drugs were similar in efficacy and toxicity, but longer-term trials are needed. Both drugs were effective.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1639855/

Record 1762

pmcid: PMC2958732

pmid:

doi:

title: Discussion.

authors

journal: British Journal of Clinical Pharmacology

journalAbbr: Br J Clin Pharmacol

year: 1976

pubdate: 1976 Feb

volume: 3

issue: 1 Suppl 1

pages: 96

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2958732/

Record 1763

pmcid: PMC1639804

pmid: 1268651

doi:

title: Editorial: Pubertal gynaecomastia.

authors

journal: British medical journal

journalAbbr: Br Med J

year: 1976

pubdate: 1976 May 22

volume: 1

issue: 6020

pages: 1238-9

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1639804/

Record 1764

pmcid: PMC1130101

pmid: 820082

doi:

title: Male hypogonadism.

authors

- Odell WD
- Swerdloff RS

journal: The Western journal of medicine

journalAbbr: West J Med

year: 1976

pubdate: 1976 Jun

volume: 124

issue: 6

pages: 446-75

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1130101/

Record 1765

pmcid: PMC1879081

pmid: 1033026

doi:

title: Metoclopramide hydrochloride and galactorrhea.

authors

- Finnis WA
- Bird CE
- Wilson DL

journal: Canadian Medical Association journal

journalAbbr: Can Med Assoc J

year: 1976

pubdate: 1976 Nov 6

volume: 115

issue: 9

pages: 845

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1879081/

Record 1766

pmcid: PMC1475237

pmid: 140799

doi: 10.1289/ehp.7617103

title: Occupational and environmental health hazards in the plastics industry.

authors

- Eckardt RE

journal: Environmental health perspectives

journalAbbr: Environ Health Perspect

year: 1976

pubdate: 1976 Oct

volume: 17

issue:

pages: 103-6

abstract: A brief description of the potential occupational health hazards encountered in the manufacture of plastics (polymers) is given. In general, these hazards are due to the monomers used to make the various plastics, to unreacted monomer contained within the finished products, and to the fillers, stabilizers, pigments, inhibitors, and initiators used in fabricating the finished products. They run the gamut from angiosarcomas of the liver due to vinyl chloride, to dermatitis and asthma-like reactions due to the initiators used with epoxy resins.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1475237/

Record 1767

pmcid: PMC1640115

pmid: 1276700

doi: 10.1136/bmj.1.6022.1385

title: Puberty: precocious and delayed.

authors

- Rayner PH

journal: British medical journal

journalAbbr: Br Med J

year: 1976

pubdate: 1976 Jun 5

volume: 1

issue: 6022

pages: 1385-7

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1640115/

Record 1768

pmcid: PMC1956052

pmid:

doi:

title: From the Overseas Journals.

authors

- Gilder SSB

journal: Canadian Medical Association Journal

journalAbbr: Can Med Assoc J

year: 1975

pubdate: 1975 Apr 19

volume: 112

issue: 8

pages: 934-40

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1956052/

Record 1769

pmcid: PMC302674

pmid:

doi: 10.1172/JCI107729

title: Abstracts.

authors

journal: Journal of Clinical Investigation

journalAbbr: J Clin Invest

year: 1974

pubdate: 1974 Jun

volume: 53

issue: 6

pages: 1a-92a

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC302674/

Record 1770

pmcid: PMC1610988

pmid:

doi:

title: Jamaica Medical Congress: 17th B.M.A. Annual Clinical Meeting, with Medical Association of Jamaica.

authors

journal: British Medical Journal

journalAbbr: Br Med J

year: 1974

pubdate: 1974 May 11

volume: 2

issue: 5914

pages: 313-23

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1610988/

Record 1771

pmcid: PMC1589234

pmid: 4699597

doi: 10.1136/bmj.2.5859.155

title: A case of hydrocolpos.

authors

- Dawson WG

journal: British medical journal

journalAbbr: Br Med J

year: 1973

pubdate: 1973 Apr 21

volume: 2

issue: 5859

pages: 155

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1589234/

Record 1772

pmcid: PMC1588487

pmid: 4689833

doi: 10.1136/bmj.1.5851.449

title: Effect of oestrogen therapy on plasma and urinary levels of uric acid.

authors

- Nicholls A
- Snaith ML
- Scott JT

journal: British medical journal

journalAbbr: Br Med J

year: 1973

pubdate: 1973 Feb 24

volume: 1

issue: 5851

pages: 449-51

abstract: Uric acid clearance studies were carried out on a low-purine diet in 22 trans-sexual men before and during oestrogen therapy for this condition (stilboestrol in 21 cases, ethinyloestradiol in one). Plasma uric acid fell in 15 of the subjects and urinary uric acid rose in 17 of 20 subjects in whom satisfactory collections were obtained. These changes are significant and it is suggested that hormonal influences are responsible for the known age and sex differences in plasma uric acid.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1588487/

Record 1773

pmcid: PMC1012975

pmid: 4572642

doi: 10.1136/jmg.10.1.50

title: Fetal and embryonic haemoglobins.

authors

- Lorkin PA

journal: Journal of medical genetics

journalAbbr: J Med Genet

year: 1973

pubdate: 1973 Mar

volume: 10

issue: 1

pages: 50-64

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1012975/

Record 1774

pmcid: PMC1013010

pmid: 4714585

doi: 10.1136/jmg.10.2.170

title: Presence of brightly fluorescent material in testes of XX males.

authors

- Palutke WA
- Chen Y
- Chen H

journal: Journal of medical genetics

journalAbbr: J Med Genet

year: 1973

pubdate: 1973 Jun

volume: 10

issue: 2

pages: 170-4

abstract: The case of a phenotypic male with a predominantly 46,XX karyotype is presented. Examination of quinacrine mustard stained frozen sections of testicular tissue revealed that 29% of Sertoli cells contained a brightly fluorescent spot, presumably representing the Y chromosome. Interestingly, these cells were the only X chromatin negative cells observed in the testis. It is concluded that it is hazardous to regard any case as a pure `XX male' without extensive study of all available tissues, including fluorescence of testicular tissue.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1013010/

Record 1775

pmcid: PMC1455080

pmid: 18730980

doi:

title: Reduction mammaplasty.

authors

- McKissock PK

journal: California medicine

journalAbbr: Calif Med

year: 1973

pubdate: 1973 Aug

volume: 119

issue: 2

pages: 65

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1455080/

Record 1776

pmcid: PMC2609013

pmid: 4735681

doi:

title: Some observations on adolescence.

authors

- Farber IJ

journal: Journal of the National Medical Association

journalAbbr: J Natl Med Assoc

year: 1973

pubdate: 1973 May

volume: 65

issue: 3

pages: 256-8

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2609013/

Record 1777

pmcid: PMC1648094

pmid: 4571396

doi: 10.1136/adc.48.3.169

title: Some physiological and clinical aspects of puberty.

authors

- Visser HK

journal: Archives of disease in childhood

journalAbbr: Arch Dis Child

year: 1973

pubdate: 1973 Mar

volume: 48

issue: 3

pages: 169-82

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1648094/

Record 1778

pmcid: PMC1931352

pmid:

doi:

title: Administrative content — journal masthead, notices, indexes, etc.

authors

journal: Can Med Assoc J

journalAbbr: Can Med Assoc J

year: 1972

pubdate: 1972 Jan 8

volume: 106

issue: 1

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1931352/

Record 1779

pmcid: PMC1787728

pmid: 5014255

doi: 10.1136/bmj.1.5803.790

title: Delayed puberty.

authors

- Price WH

journal: British medical journal

journalAbbr: Br Med J

year: 1972

pubdate: 1972 Mar 25

volume: 1

issue: 5803

pages: 790-3

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1787728/

Record 1780

pmcid: PMC302176

pmid: 4622109

doi: 10.1172/JCI106859

title: Functional evaluation of prolactin secretion: a guide to therapy.

authors

- Friesen H
- Guyda H
- Hwang P
- Tyson JE
- Barbeau A

journal: The Journal of clinical investigation

journalAbbr: J Clin Invest

year: 1972

pubdate: 1972 Mar

volume: 51

issue: 3

pages: 706-9

abstract: Stimulation and inhibition tests are proposed for evaluating prolactin secretion. Thyrotropin-releasing hormone (TRH) stimulates the release of prolactin from the pituitary. Chlorpromazine acts presumably at the hypothalamic level to increase prolactin secretion. L-Dopa (D,L-alpha-hydrazino-alpha-methyl-beta-[3,4-di-hydroxyphenyl]) has the opposite effect; it inhibits prolactin secretion and may be effective in suppressing galactorrhea.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC302176/

Record 1781

pmcid: PMC2495298

pmid: 18557246

doi: 10.1136/pgmj.48.559.317

title: Precocious sexual development in an untreated cretin.

authors

- Bulugahapitiya DT

journal: Postgraduate medical journal

journalAbbr: Postgrad Med J

year: 1972

pubdate: 1972 May

volume: 48

issue: 559

pages: 317-20

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2495298/

Record 1782

pmcid: PMC302199

pmid: 4111366

doi: 10.1172/JCI106880

title: Study of four new kindreds with inherited thyroxine-binding globulin abnormalities. Possible mutations of a single gene locus.

authors

- Refetoff S
- Robin NI
- Alper CA

journal: The Journal of clinical investigation

journalAbbr: J Clin Invest

year: 1972

pubdate: 1972 Apr

volume: 51

issue: 4

pages: 848-67

abstract: Five families with inherited thyroxine-binding globulin (TBG) abnormalities were studied. On the basis of serum thyroxine (T(4))- binding capacity of TBG in affected males, three family types were identified: TBG deficiency, low TBG, and high TBG capacity. In all families evidence for X-linked inheritance was obtained and in one family all criteria establishing this mode of inheritance were met. Only females were heterozygous, exhibiting values intermediate between affected males and normals. Overlap in heterozygotes was most commonly encountered in families with low TBG. QUANTITATIVE VARIATION IN THE SERUM CONCENTRATION OF FUNCTIONALLY NORMAL TBG WAS DEMONSTRATED BY: (a) failure of serum from TBG-deficient subjects to react with anti-TBG antibodies; (b) normal kinetics of T(4) and triiodothyronine-binding to TBG in sera from subjects with low TBG and high TBG capacity; (c) concordance of estimates of TBG concentration by T(4) saturation and by immunological methods; and (d) normal rate of heat inactivation of TBG. No abnormalities in serum transport of cortisol, testosterone, aldosterone, or thyroxine bound to prealbumin

could be detected. These observations suggest that all the TBG abnormalities thus far observed reflect mutations at a single X-linked locus involved in the control of TBG synthesis.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC302199/

Record 1783

pmcid: PMC1647957

pmid: 5018626

doi: 10.1136/adc.47.251.119

title: Syndrome of growth resistance, obesity, and intellectual impairment with precocious puberty.

authors

- Macmillan DR
- Kim CB
- Weisskopf B

journal: Archives of disease in childhood

journalAbbr: Arch Dis Child

year: 1972

pubdate: 1972 Feb

volume: 47

issue: 251

pages: 119-21

abstract: A syndrome of growth resistance, obesity, and intellectual impairment with precocious puberty is described. Two girls presenting with many of the manifestations of the Prader-Willi syndrome, including growth resistance, obesity, and intellectual impairment, also exhibited precocious sexual maturation. It is suggested that this syndrome represents a hypothalamic disturbance closely related to the Prader-Willi syndrome and should be considered one of its variants.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1647957/

Record 1784

pmcid: PMC1931098

pmid:

doi:

title: Administrative content — journal masthead, notices, indexes, etc.

authors

journal: Can Med Assoc J

journalAbbr: Can Med Assoc J

year: 1971

pubdate: 1971 Aug 21

volume: 105

issue: 4

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1931098/

Record 1785

pmcid: PMC1931397

pmid:

doi:

title: Administrative content — journal masthead, notices, indexes, etc.

authors

journal: Can Med Assoc J

journalAbbr: Can Med Assoc J

year: 1971

pubdate: 1971 Dec 4

volume: 105

issue: 11

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1931397/

Record 1786

pmcid: PMC1930976

pmid:

doi:

title: Administrative content — journal masthead, notices, indexes, etc.

authors

journal: Can Med Assoc J

journalAbbr: Can Med Assoc J

year: 1971

pubdate: 1971 Apr 3

volume: 104

issue: 7

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1930976/

Record 1787

pmcid: PMC1647569

pmid: 5555493

doi: 10.1136/adc.46.245.91

title: Polyostotic fibrous dysplasia with raised steroid excretion.

authors

- Alexander FW

  journal: Archives of disease in childhood

  journalAbbr: Arch Dis Child

  year: 1971

  pubdate: 1971 Feb

  volume: 46

  issue: 245

  pages: 91-4

  abstract: Four female patients with polyostotic fibrous dysplasia and sexual precocity are presented in detail. Two of the patients suffered spontaneous fracture of the femur. Steroid excretion was raised in 3 out of the 4 girls, but excretion of total urinary gonadotrophins was normal in 2 of them.

  url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1647569/

Record 1788

pmcid: PMC2608629

pmid: 5121158

doi:

title: The surgical enigma of gynecomastia.

authors

- Walker WW

  journal: Journal of the National Medical Association

  journalAbbr: J Natl Med Assoc

  year: 1971

  pubdate: 1971 Sep

  volume: 63

  issue: 5

  pages: 385-6

  abstract:

  url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2608629/

Record 1789

pmcid: PMC1930510

pmid:

doi:

title: Administrative content — journal masthead, notices, indexes, etc.

authors

journal: Can Med Assoc J

journalAbbr: Can Med Assoc J

year: 1970

pubdate: 1970 Sep 26

volume: 103

issue: 6

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1930510/

Record 1790

pmcid: PMC322673

pmid: 5460287

doi: 10.1172/JCI106402

title: Correlation of serum follicular stimulating hormone (FSH) and luteinizing hormone (LH) as measured by radioimmunoassay in disorders of sexual development.

authors

- Penny R
- Guyda HJ
- Baghdassarian A
- Johanson AJ
- Blizzard RM

journal: The Journal of clinical investigation

journalAbbr: J Clin Invest

year: 1970

pubdate: 1970 Oct

volume: 49

issue: 10

pages: 1847-52

abstract: Serum FSH and LH levels in 104 patients with disorders of sexual development were determined by radioimmunoassay and compared with serum FSH and LH levels in 164 normal individuals.32 of 35 gonadal dysgenesis patients (ages 4.8-18.9 yr) had serum FSH levels which were elevated above the range of normal for chronological age, and 19 had serum LH levels similarly elevated. All patients with elevated serum LH levels were 11 yr of age or older. However, 8 of 10 gonadal dysgenesis patients, ages 4.8-10.9 yr, had serum FSH levels elevated above the normal range. In accord with these observations was the finding that in normal girls, serum FSH levels may increase at an earlier age than do serum LH levels (FSH, 5-8 yr of age; LH, 9-10 yr of age). These data indicate that serum FSH determinations may be helpful in diagnosing gonadal dysgenesis during childhood. Serum gonadotropin levels within the range of normal for chronological age were found in 2 of 18 girls with idiopathic isosexual precocity. The other 16 had serum FSH levels elevated above the range of normal for chronological age, and 8 also had serum LH levels similarly elevated. In all instances serum FSH and LH levels were in the range expected for the stage of sexual development. In 35 boys, ages 13.1-17.8 yr, with delayed adolescence, serum gonadotropin levels correlated with stage of sexual development and, therefore, were often less than those expected for age.8 patients with premature pubarche, 5 patients with premature thelarche, and 3 patients with adolescent gynecomastia had serum gonadotropin levels within the range of normal for chronological age.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC322673/

Record 1791

pmcid: PMC1501582

pmid: 5457518

doi:

title: Hemangiomatosis of the liver with thrombosis following use of an oral contraceptive.

authors

- Kositchek RJ
- Cullen RA

journal: California medicine

journalAbbr: Calif Med

year: 1970

pubdate: 1970 Sep

volume: 113

issue: 3

pages: 70-4

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1501582/

Record 1792

pmcid: PMC1946692

pmid: 20311607

doi:

title: Yesterday'S medicine: the genesis, classification, interrelationships and clinical diagnosis of disease.

authors

- Anderson HB

journal: Canadian Medical Association journal

journalAbbr: Can Med Assoc J

year: 1970

pubdate: 1970 Apr 25

volume: 102

issue: 8

pages: 793

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1946692/

Record 1793

pmcid: PMC1810802

pmid:

doi:

title: Administrative content — journal masthead, notices, indexes, etc.

authors

journal: Proc R Soc Med

journalAbbr: Proc R Soc Med

year: 1969

pubdate: 1969 Oct

volume: 62

issue: 10

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1810802/

Record 1794

pmcid: PMC1983427

pmid:

doi:

title: Any Questions?

authors

journal: British Medical Journal

journalAbbr: Br Med J

year: 1969

pubdate: 1969 May 24

volume: 2

issue: 5655

pages: 498

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1983427/

Record 1795

pmcid: PMC2611606

pmid: 5763313

doi:

title: Atypical cerebral gigantism. A case report.

authors

- Jung JS
- Martin BF

journal: Journal of the National Medical Association

journalAbbr: J Natl Med Assoc

year: 1969

pubdate: 1969 Jan

volume: 61

issue: 1

pages: 44-6

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2611606/

Record 1796

pmcid: PMC1945705

pmid: 5765409

doi:

title: Gynecomastia during hemodialysis.

authors

journal: Canadian Medical Association journal

journalAbbr: Can Med Assoc J

year: 1969

pubdate: 1969 Feb 22

volume: 100

issue: 8

pages: 394-5

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1945705/

Record 1797

pmcid: PMC1629889

pmid: 5388733

doi: 10.1136/bmj.4.5677.236-a

title: Inappropriate lactation.

authors

- Segal M
- Heys RF

journal: British medical journal

journalAbbr: Br Med J

year: 1969

pubdate: 1969 Oct 25

volume: 4

issue: 5677

pages: 236

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1629889/

Record 1798

pmcid: PMC1226736

pmid: 5388019

doi: 10.2105/ajph.59.10.1910

title: The value of periodic school health examinations.

authors

- Meyerstein AN

journal: American journal of public health and the nation's health

journalAbbr: Am J Public Health Nations Health

year: 1969

pubdate: 1969 Oct

volume: 59

issue: 10

pages: 1910-26

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1226736/

Record 1799

pmcid: PMC1984874

pmid: 5638272

doi: 10.1136/bmj.1.5588.381-c

title: Gynaecomastia in renal failure.

authors

- Freeman RM
- Lawton RL
- Fearing MO

journal: British medical journal

journalAbbr: Br Med J

year: 1968

pubdate: 1968 Feb 10

volume: 1

issue: 5588

pages: 381-2

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1984874/

Record 1800

pmcid: PMC1503026

pmid: 18730098

doi:

title: Gynecomastia, hypospermatogenesis and a large y chromosome in a man.

authors

- Dumars KW
- Fisher CE

journal: California medicine

journalAbbr: Calif Med

year: 1968

pubdate: 1968 Jan

volume: 108

issue: 1

pages: 52-4

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1503026/

Record 1801

pmcid: PMC1749077

pmid: 6081005

doi: 10.1136/bmj.4.5582.779

title: Gynaecomastia in chronic renal failure.

authors

- Lindsay RM
- Briggs JD
- Luke RG
- Boyle IT
- Kennedy AC

journal: British medical journal

journalAbbr: Br Med J

year: 1967

pubdate: 1967 Dec 30

volume: 4

issue: 5582

pages: 779-80

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1749077/

Record 1802

pmcid: PMC1502785

pmid: 4863186

doi:

title: Gynecomastia associated with vincristine therapy.

authors

- Smith RH
- Barrett O Jr

journal: California medicine

journalAbbr: Calif Med

year: 1967

pubdate: 1967 Oct

volume: 107

issue: 4

pages: 347-9

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1502785/

Record 1803

pmcid: PMC1923508

pmid:

doi:

title: Medical News in Brief.

authors

journal: Canadian Medical Association Journal

journalAbbr: Can Med Assoc J

year: 1967

pubdate: 1967 Nov 18

volume: 97

issue: 21

pages: 44-50

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1923508/

Record 1804

pmcid: PMC1923675

pmid:

doi:

title: Medical News in Brief.

authors

journal: Canadian Medical Association Journal

journalAbbr: Can Med Assoc J

year: 1967

pubdate: 1967 Dec 9

volume: 97

issue: 24

pages: 29-41

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1923675/

Record 1805

pmcid: PMC1944654

pmid:

doi:

title: Burkitt's Tumour in Pregnancy.

authors

- Bannerman RHO

journal: British Medical Journal

journalAbbr: Br Med J

year: 1966

pubdate: 1966 Nov 5

volume: 2

issue: 5522

pages: 1136-7

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1944654/

Record 1806

pmcid: PMC2165459

pmid:

doi:

title: Leprosy and Gynaecomastia.

authors

- Murley AHG

journal: British Medical Journal

journalAbbr: Br Med J

year: 1965

pubdate: 1965 Feb 13

volume: 1

issue: 5432

pages: 451

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2165459/

Record 1807

pmcid: PMC1477007

pmid: 5845590

doi: 10.1097/00000658-196512000-00017

title: Mastoplasty for massive bilateral benign breast hypertrophy associated with pregnancy: case report.

authors

- Greeley PW
- Robertson LE
- Curtin JW

journal: Annals of surgery

journalAbbr: Ann Surg

year: 1965

pubdate: 1965 Dec

volume: 162

issue: 6

pages: 1081-3

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1477007/

Record 1808

pmcid: PMC1408506

pmid: 14119193

doi: 10.1097/00000658-196402000-00013

title: ADRENAL GIANT CYST: HEMANGIOMA OF MEDULLA WITH OSMOTIC PSEUDOCYST FORMATION.

authors

- ELLIOTT GB
- WALKER RH
- WRIGHT AS
- ELLIOTT KA

journal: Annals of surgery

journalAbbr: Ann Surg

year: 1964

pubdate: 1964 Feb

volume: 159

issue: 2

pages: 275-8

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1408506/

Record 1809

pmcid: PMC2019153

pmid: 14160089

doi: 10.1136/adc.39.203.66

title: FOUR CLINICAL VARIANTS OF CONGENITAL ADRENAL HYPERPLASIA.

authors

- HAMILTON W
- BRUSH MG

journal: Archives of disease in childhood

journalAbbr: Arch Dis Child

year: 1964

pubdate: 1964 Feb

volume: 39

issue: 203

pages: 66-72

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2019153/

Record 1810

pmcid: PMC2610889

pmid: 14114982

doi:

title: FUNCTIONAL OVARIAN TUMORS: A TEN YEAR STUDY AT FREEDMEN'S HOSPITAL.

authors

- HILL WE
- CLARK JF

journal: Journal of the National Medical Association

journalAbbr: J Natl Med Assoc

year: 1964

pubdate: 1964 Jan

volume: 56

issue: 1

pages: 66-70

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2610889/

Record 1811

pmcid: PMC1408831

pmid: 14236608

doi: 10.1097/00000658-196411000-00014

title: PATHOLOGIC ANATOMY IN CUSHING'S SYNDROME.

authors

- O'NEAL LW

journal: Annals of surgery

journalAbbr: Ann Surg

year: 1964

pubdate: 1964 Nov

volume: 160

issue: 5

pages: 860-9

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1408831/

Record 1812

pmcid: PMC2124093

pmid: 13942986

doi: 10.1136/bmj.1.5343.1460

title: Lactation due to methyldopa.

authors

- PETTINGER WA
- HORWITZ D
- SJOERDSMA A

journal: British medical journal

journalAbbr: Br Med J

year: 1963

pubdate: 1963 Jun 1

volume: 1

issue: 5343

pages: 1460

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2124093/

Record 1813

pmcid: PMC1897095

pmid: 13944759

doi: 10.1177/003591576305600206

title: Some aspects of the conservative management of prostatic cancer.

authors

- FERGUSSON JD

journal: Proceedings of the Royal Society of Medicine

journalAbbr: Proc R Soc Med

year: 1963

pubdate: 1963 Feb

volume: 56

issue: 2

pages: 81-8

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1897095/

Record 1814

pmcid: PMC1254174

pmid: 13992391

doi: 10.2105/ajph.53.6.890

title: Some observations on the epidemiology of breast cancer among males.

authors

- SCHOTTENFELD D
- LILIENFELD AM
- DIAMOND H

journal: American journal of public health and the nation's health

journalAbbr: Am J Public Health Nations Health

year: 1963

pubdate: 1963 Jun

volume: 53

issue: 6

pages: 890-7

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1254174/

Record 1815

pmcid: PMC1466413

pmid: 13997865

doi: 10.1097/00000658-196301000-00016

title: Surgical treatment of gynecomastia: an analysis of 275 cases.

authors

- VON KESSEL F
- PICKRELL KL
- HUGER WE
- MATTON G

journal: Annals of surgery

journalAbbr: Ann Surg

year: 1963

pubdate: 1963 Jan

volume: 157

issue: 1

pages: 142-51

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1466413/

Record 1816

pmcid: PMC1959177

pmid:

doi:

title: ANY QUESTIONS?

authors

journal: British Medical Journal

journalAbbr: Br Med J

year: 1962

pubdate: 1962 Jun 30

volume: 1

issue: 5295

pages: 1845-6

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1959177/

Record 1817

pmcid: PMC1926558

pmid: 14033615

doi: 10.1136/bmj.2.5313.1157

title: Effect of oestrogens on myometrial contractions.

authors

- KLOPPER AI
- DENNIS KJ

journal: British medical journal

journalAbbr: Br Med J

year: 1962

pubdate: 1962 Nov 3

volume: 2

issue: 5313

pages: 1157-9

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1926558/

Record 1818

pmcid: PMC1466100

pmid: 13915699

doi: 10.1097/00000658-196204000-00013

title: Hydrometrocolpos: report of three cases and review of the literature.

authors

- SPENCER R
- LEVY DM

journal: Annals of surgery

journalAbbr: Ann Surg

year: 1962

pubdate: 1962 Apr

volume: 155

issue: 4

pages: 558-71

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1466100/

Record 1819

pmcid: PMC2482156

pmid: 13944426

doi: 10.1136/pgmj.38.445.612

title: Maternal hyperglycaemia and foetal hyperinsulinism in diabetic pregnancy.

authors

- FARQUHAR JW

journal: Postgraduate medical journal

journalAbbr: Postgrad Med J

year: 1962

pubdate: 1962 Nov

volume: 38

issue: 445

pages: 612-28

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2482156/

Record 1820

pmcid: PMC1957589

pmid: 13863811

doi: 10.1136/bmj.1.5275.369

title: Schistosomiasis of the cervix.

authors

- BADAWY AH

journal: British medical journal

journalAbbr: Br Med J

year: 1962

pubdate: 1962 Feb 10

volume: 1

issue: 5275

pages: 369-72

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1957589/

Record 1821

pmcid: PMC1574854

pmid: 13885463

doi:

title: The thorny child.

authors

- DENNIS JL

journal: California medicine

journalAbbr: Calif Med

year: 1962

pubdate: 1962 Feb

volume: 96

issue: 2

pages: 89-93

abstract: Too many physicians-and parents-hide behind the overworked excuse that "Johnny is just going through a stage." If the remark is inaccurate a great disservice can be done to both mother and child, and ultimately to society. The well oriented physician would no more permit a young mother to unwittingly feel "guilty" because her two-year-old "little stinker" behaves like a two-year-old little stinker than he would casually reassure when a ten-year-old behaves as though he were two. Actually much of the unpleasant behavior of children is quite normal. If physicians would help all young mothers to recognize this without dismissing abnormal behavior, it would do much to avert the overwhelming sense of inadequacy that so many modern young mothers feel-especially with their first baby. If they can be made comfortable with their first the others usually come easily. Many physicians who care for children are not trained in the rudiments of developmental behavior. By means of a simple outline and drawing of "the thorny child" even the least of the experts can better understand some of the chronologic variations in developmental behavior.

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1574854/

Record 1822

pmcid: PMC1939369

pmid:

doi:

title: Abstracts.

authors

journal: Canadian Medical Association Journal

journalAbbr: Can Med Assoc J

year: 1961

pubdate: 1961 Mar 18

volume: 84

issue: 11

pages: 11-6

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1939369/

Record 1823

pmcid: PMC1939348

pmid: 13753182

doi:

title: The use of norethynodrel (enovid) in clinical practice.

authors

- KORN GW

journal: Canadian Medical Association journal

journalAbbr: Can Med Assoc J

year: 1961

pubdate: 1961 Mar 18

volume: 84

issue: 11

pages: 584-7

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1939348/

Record 1824

pmcid: PMC1578316

pmid:

doi:

title: Administrative content — journal masthead, notices, indexes, etc.

authors

journal: Calif Med

journalAbbr: Calif Med

year: 1960

pubdate: 1960 Sep

volume: 93

issue: 3

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1578316/

Record 1825

pmcid: PMC2098249

pmid: 13723144

doi: 10.1136/bmj.2.5206.1103

title: Advanced extrauterine pregnancy.

authors

- DIXON HG
- STEWART DB

journal: British medical journal

journalAbbr: Br Med J

year: 1960

pubdate: 1960 Oct 15

volume: 2

issue: 5206

pages: 1103-11

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2098249/

Record 1826

pmcid: PMC2012613

pmid: 13824046

doi: 10.1136/adc.35.181.296

title: Calcification of the digital vessels in a child with rheumatoid arthritis.

authors

- FORSYTH CC

journal: Archives of disease in childhood

journalAbbr: Arch Dis Child

year: 1960

pubdate: 1960 Jun

volume: 35

issue: 181

pages: 296-301

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2012613/

Record 1827

pmcid: PMC293334

pmid: 14442011

doi: 10.1172/JCI104067

title: Clinical, morphological and biochemical studies of a virilizing tumor in the testis.

authors

- SAVARD K
- DORFMAN RI
- BAGGETT B
- FIELDING LL
- ENGEL LL
- McPHERSON HT
- LISTER LM
- JOHNSON DS
- HAMBLEN EC
- ENGEL FL

journal: The Journal of clinical investigation

journalAbbr: J Clin Invest

year: 1960

pubdate: 1960 Mar

volume: 39

issue: 3

pages: 534-53

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC293334/

Record 1828

pmcid: PMC1613666

pmid: 17859631

doi:

title: Investigation of, and Use of DiMethyl Siloxanes, Halogenated Carbons and Polyvinyl Alcohol, as Subcutaneous Prosthesis.

authors

- Brown JB
- Ohlwiler DA
- Fryer MP

journal: Annals of surgery

journalAbbr: Ann Surg

year: 1960

pubdate: 1960 Sep

volume: 152

issue: 3

pages: 534-47

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1613666/

Record 1829

pmcid: PMC1613606

pmid: 13846227

doi: 10.1097/00000658-196007000-00021

title: Large breast tumors in adolescent females.

authors

- WULSIN JH

journal: Annals of surgery

journalAbbr: Ann Surg

year: 1960

pubdate: 1960 Jul

volume: 152

issue: 1

pages: 151-9

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1613606/

Record 1830

pmcid: PMC1937639

pmid: 14446347

doi:

title: Should spontaneous abortion be prevented? Vitamin E in its management.

authors

- SHUTE EV

journal: Canadian Medical Association journal

journalAbbr: Can Med Assoc J

year: 1960

pubdate: 1960 Jan 9

volume: 82

issue: 2

pages: 72-6

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1937639/

Record 1831

pmcid: PMC2012346

pmid: 13650663

doi: 10.1136/adc.34.174.120

title: A case of calcification of the basal ganglia with osteoporosis and other abnormalities.

authors

- BENSON PF

journal: Archives of disease in childhood

journalAbbr: Arch Dis Child

year: 1959

pubdate: 1959 Apr

volume: 34

issue: 174

pages: 120-4

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2012346/

Record 1832

pmcid: PMC1577631

pmid:

doi:

title: Administrative content — journal masthead, notices, indexes, etc.

authors

journal: Calif Med

journalAbbr: Calif Med

year: 1959

pubdate: 1959 Apr

volume: 90

issue: 4

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1577631/

Record 1833

pmcid: PMC2012437

pmid: 13840819

doi: 10.1136/adc.34.178.495

title: Masculinization of a female foetus with oestrogenic effect.

authors

- VALENTINE GH

journal: Archives of disease in childhood

journalAbbr: Arch Dis Child

year: 1959

pubdate: 1959 Dec

volume: 34

issue: 178

pages: 495-7

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2012437/

Record 1834

pmcid: PMC1870605

pmid: 13645665

doi: 10.1177/003591575905200422

title: Non-adrenal female pseudohermaphroditism associated with hormone administration in pregnancy.

authors

- JOLLY H

journal: Proceedings of the Royal Society of Medicine

journalAbbr: Proc R Soc Med

year: 1959

pubdate: 1959 Apr

volume: 52

issue: 4

pages: 300-1

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1870605/

Record 1835

pmcid: PMC2012187

pmid: 13509743

doi: 10.1136/adc.33.167.49

title: Acromegaly in a girl of 8 years.

authors

- TODD RM

journal: Archives of disease in childhood

journalAbbr: Arch Dis Child

year: 1958

pubdate: 1958 Feb

volume: 33

issue: 167

pages: 49-54

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2012187/

Record 1836

pmcid: PMC2012324

pmid: 13584028

doi: 10.1136/adc.33.171.455

title: The prevalence of dental caries in relation to maturity.

authors

- MANSBRIDGE JN

journal: Archives of disease in childhood

journalAbbr: Arch Dis Child

year: 1958

pubdate: 1958 Oct

volume: 33

issue: 171

pages: 455-64

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2012324/

Record 1837

pmcid: PMC1464866

pmid: 13292880

doi: 10.1097/00000658-195602000-00012

title: Adrenalin producing tumors in childhood.

authors

- MOORE TC
- SHUMACKER HB Jr

journal: Annals of surgery

journalAbbr: Ann Surg

year: 1956

pubdate: 1956 Feb

volume: 143

issue: 2

pages: 256-65

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1464866/

Record 1838

pmcid: PMC1465124

pmid: 13303096

doi: 10.1097/00000658-195604000-00019

title: Bleeding from the nipple in infancy due to cystic ductal hyperplasia of the breast.

authors

- MYERS MB

- KAPLAN IW

journal: Annals of surgery

journalAbbr: Ann Surg

year: 1956

pubdate: 1956 Apr

volume: 143

issue: 4

pages: 557-60

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1465124/

Record 1839

pmcid: PMC2501204

pmid: 13389020

doi: 10.1136/pgmj.32.373.550

title: Ectopic pregnancy.

authors

- PERCIVAL RC

journal: Postgraduate medical journal

journalAbbr: Postgrad Med J

year: 1956

pubdate: 1956 Nov

volume: 32

issue: 373

pages: 550-3; passim

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2501204/

Record 1840

pmcid: PMC2012004

pmid: 13395578

doi: 10.1136/adc.31.160.483

title: The possible influence of hyperadrenocorticism on the foetus of the diabetic woman.

authors

- FARQUHAR JW

journal: Archives of disease in childhood

journalAbbr: Arch Dis Child

year: 1956

pubdate: 1956 Dec

volume: 31

issue: 160

pages: 483-501

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2012004/

Record 1841

pmcid: PMC1825388

pmid:

doi:

title: Abstracts.

authors

journal: Canadian Medical Association Journal

journalAbbr: Can Med Assoc J

year: 1955

pubdate: 1955 Jan 1

volume: 72

issue: 1

pages: 59-63

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1825388/

Record 1842

pmcid: PMC2011793

pmid: 13249619

doi: 10.1136/adc.30.152.328

title: An anthropometric study of Edinburgh schoolchildren. I. Methods, data and assessment of maturity.

authors

- PROVIS HS
- ELLIS RW

journal: Archives of disease in childhood

journalAbbr: Arch Dis Child

year: 1955

pubdate: 1955 Aug

volume: 30

issue: 152

pages: 328-37

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2011793/

Record 1843

pmcid: PMC1980778

pmid: 13250194

doi: 10.1136/bmj.2.4941.712

title: Eunuchoidism in identical twins; with congenital fusion of the thoracic vertebrae.

authors

- ELLENBOGEN BK
- RYAN GM

journal: British medical journal

journalAbbr: Br Med J

year: 1955

pubdate: 1955 Sep 17

volume: 2

issue: 4941

pages: 712-5

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1980778/

Record 1844

pmcid: PMC2093641

pmid: 13149869

doi: 10.1136/bmj.1.4867.912

title: Clinical and chemical studies in human lactation. VI. The functional capacity of the breast.

authors

- HYTTEN FE

journal: British medical journal

journalAbbr: Br Med J

year: 1954

pubdate: 1954 Apr 17

volume: 1

issue: 4867

pages: 912-5

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2093641/

Record 1845

pmcid: PMC2079465

pmid: 13190231

doi: 10.1136/bmj.2.4889.685

title: Gynaecomastis in two brothers.

authors

- FERRIMAN DG

journal: British medical journal

journalAbbr: Br Med J

year: 1954

pubdate: 1954 Sep 18

volume: 2

issue: 4889

pages: 685-6

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2079465/

Record 1846

pmcid: PMC1824858

pmid:

doi:

title: Book Reviews.

authors

journal: Canadian Medical Association Journal

journalAbbr: Can Med Assoc J

year: 1953

pubdate: 1953 Nov

volume: 69

issue: 5

pages: 557-66

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1824858/

Record 1847

pmcid: PMC1987531

pmid: 13003950

doi: 10.1177/003591575204500905

title: Isosexual precocity.

authors

- LEYS D

journal: Proceedings of the Royal Society of Medicine

journalAbbr: Proc R Soc Med

year: 1952

pubdate: 1952 Sep

volume: 45

issue: 9

pages: 590-1

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1987531/

Record 1848

pmcid: PMC1988756

pmid: 14924692

doi: 10.1136/adc.27.132.200

title: PART III. Abnormalities of the breast.

authors

journal: Archives of disease in childhood

journalAbbr: Arch Dis Child

year: 1952

pubdate: 1952 Apr

volume: 27

issue: 132

pages: 200-4

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1988756/

Record 1849

pmcid: PMC2081798

pmid: 14864525

doi: 10.1177/003591575104400611

title: Adipose gynandrism and gynism.

authors

- SIMPSON SL

journal: Proceedings of the Royal Society of Medicine

journalAbbr: Proc R Soc Med

year: 1951

pubdate: 1951 Jun

volume: 44

issue: 6

pages: 453-9

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2081798/

Record 1850

pmcid: PMC2081780

pmid: 14864526

doi: 10.1177/003591575104400612

title: Sexual precocity.

authors

- JOLLY H

journal: Proceedings of the Royal Society of Medicine

journalAbbr: Proc R Soc Med

year: 1951

pubdate: 1951 Jun

volume: 44

issue: 6

pages: 459-63

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2081780/

Record 1851

pmcid: PMC2184657

pmid: 18864704

doi: 10.1177/003591574804100529

title: Acne conglobata treated by implantation of 300 mg. of stilboestrol.

authors

- FORMAN L

journal: Proceedings of the Royal Society of Medicine

journalAbbr: Proc R Soc Med

year: 1948

pubdate: 1948 May

volume: 41

issue: 5

pages: 300

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2184657/

Record 1852

pmcid: PMC2091669

pmid: 18882986

doi: 10.1136/bmj.2.4576.555

title: Gynatresia; report of three uncommon clinical types.

authors

- MALIPHANT RG

journal: British medical journal

journalAbbr: Br Med J

year: 1948

pubdate: 1948 Sep 18

volume: 2

issue: 4576

pages: 555-8

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2091669/

Record 1853

pmcid: PMC2598785

pmid: 18103721

doi:

title: Nature and significance of neutral steroids in human urine in normal and in abnormal states; with a preliminary consideration of the adrenal and gonadal steroids and the factors which influence their secretion and biological action.

authors

- ENGSTROM WW

journal: The Yale journal of biology and medicine

journalAbbr: Yale J Biol Med

year: 1948

pubdate: 1948 Oct

volume: 21

issue: 1

pages: 21-85

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2598785/

Record 1854

pmcid: PMC2184538

pmid: 18864722

doi: 10.1177/003591574804100604

title: Obesity, hypogonadism, hypothyrokinesia, adenoma of thyroid gland and failure of growth in a girl aged 13 years.

authors

- GREENE R

journal: Proceedings of the Royal Society of Medicine

journalAbbr: Proc R Soc Med

year: 1948

pubdate: 1948 Jun

volume: 41

issue: 6

pages: 352

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2184538/

Record 1855

pmcid: PMC1942774

pmid: 18894822

doi:

title: Paragangliomas; review of subject and report of five original cases.

authors

- BRINES OA
- JENNINGS ER

journal: The American journal of pathology

journalAbbr: Am J Pathol

year: 1948

pubdate: 1948 Nov

volume: 24

issue: 6

pages: 1167-97

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1942774/

Record 1856

pmcid: PMC1871474

pmid: 18860462

doi:

title: Testicular dysfunction.

authors

- McCULLAGH EP

journal: Bulletin of the New York Academy of Medicine

journalAbbr: Bull N Y Acad Med

year: 1948

pubdate: 1948 Jun

volume: 24

issue: 6

pages: 341-63

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1871474/

Record 1857

pmcid: PMC2055111

pmid:

doi:

title: QUESTIONS AND COMMENTS.

authors

journal: British Medical Journal

journalAbbr: Br Med J

year: 1947

pubdate: 1947 Jul 5

volume: 2

issue: 4513

pages: 40-2

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2055111/

Record 1858

pmcid: PMC1988060

pmid: 20267736

doi: 10.1136/adc.22.111.152

title: The excretion of 17-ketosteroids and gonadotrophin in children: normal and abnormal cases.

authors

- HAIN AM

journal: Archives of disease in childhood

journalAbbr: Arch Dis Child

year: 1947

pubdate: 1947 Sep

volume: 22

issue: 111

pages: 152-8

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1988060/

Record 1859

pmcid: PMC1987986

pmid: 20297374

doi: 10.1136/adc.21.108.181

title: Height and weight in relation to onset of puberty in boys.

authors

- ELLIS RW

journal: Archives of disease in childhood

journalAbbr: Arch Dis Child

year: 1946

pubdate: 1946 Dec

volume: 21

issue: 108

pages: 181-9

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1987986/

Record 1860

pmcid: PMC1827525

pmid: 20322605

doi:

title: Lactation in Women.

authors

- Gunther M

journal: Canadian Medical Association journal

journalAbbr: Can Med Assoc J

year: 1942

pubdate: 1942 Nov

volume: 47

issue: 5

pages: 410-4

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1827525/

Record 1861

pmcid: PMC2160517

pmid: 20783497

doi: 10.1136/bmj.1.4177.117

title: Feminism.

authors

- Broster LR

journal: British medical journal

journalAbbr: Br Med J

year: 1941

pubdate: 1941 Jan 25

volume: 1

issue: 4177

pages: 117-126.2

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2160517/

Record 1862

pmcid: PMC1391282

pmid: 17857378

doi: 10.1097/00000658-193906000-00002

title: BENIGN AND MALIGNANT LESIONS OF THE MALE BREAST.

authors

- Horsley JS

journal: Annals of surgery

journalAbbr: Ann Surg

year: 1939

pubdate: 1939 Jun

volume: 109

issue: 6

pages: 912-20

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1391282/

Record 1863

pmcid: PMC2602231

pmid: 21433876

doi:

title: The Clinical Use of Stilbestrol, A Synthetic Estrogen: Preliminary Report.

authors

- Lewis RM

journal: The Yale journal of biology and medicine

journalAbbr: Yale J Biol Med

year: 1939

pubdate: 1939 Dec

volume: 12

issue: 2

pages: 235-8

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2602231/

Record 1864

pmcid: PMC2087743

pmid: 20781050

doi:

title: Annotations.

authors

journal: British medical journal

journalAbbr: Br Med J

year: 1937

pubdate: 1937 Nov 13

volume: 2

issue: 4010

pages: 973-5

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2087743/

Record 1865

pmcid: PMC424908

pmid: 16694515

doi: 10.1172/JCI100895

title: THE URINARY EXCRETION OF ANDROGENIC AND ESTROGENIC SUBSTANCES IN CERTAIN ENDOCRINE STATES. STUDIES IN HYPOGONADISM, GYNECOMASTIA AND VIRILISM.

authors

- Kenyon AT
- Gallagher TF
- Peterson DH
- Dorfman RI
- Koch FC

journal: The Journal of clinical investigation

journalAbbr: J Clin Invest

year: 1937

pubdate: 1937 Sep

volume: 16

issue: 5

pages: 705-17

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC424908/

Record 1866

pmcid: PMC5424060

pmid: 29612419

doi:

title: Exhibition of Specimens.

authors

journal: Transactions. Edinburgh Obstetrical Society

journalAbbr: Trans Edinb Obstet Soc

year: 1929

pubdate: 1929

volume: 49

issue:

pages: 209-210

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5424060/

Record 1867

pmcid: PMC1399975

pmid: 17865246

doi: 10.1097/00000658-192504000-00017

title: TRANSACTIONS OF THE NEW YORK SURGICAL SOCIETY 10, 1924: Stated Meeting Held December.

authors

journal: Annals of surgery

journalAbbr: Ann Surg

year: 1925

pubdate: 1925 Apr

volume: 81

issue: 4

pages: 869-75

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1399975/

Record 1868

pmcid: PMC2316634

pmid: 20771122

doi: 10.1136/bmj.1.3253.747

title: A Lecture ON CANCER OF THE BREAST.

authors

- Rowntree C

journal: British medical journal

journalAbbr: Br Med J

year: 1923

pubdate: 1923 May 5

volume: 1

issue: 3253

pages: 747-9

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2316634/

Record 1869

pmcid: PMC2414336

pmid:

doi:

title: Administrative content — journal masthead, notices, indexes, etc.

authors

journal: Br Med J

journalAbbr: Br Med J

year: 1921

pubdate: 1921 Jan 1

volume: 1

issue: 3131

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2414336/

Record 1870

pmcid: PMC2414624

pmid:

doi:

title: EPITOME OF CURRENT MEDICAL LITERATURE.

authors

journal: British Medical Journal

journalAbbr: Br Med J

year: 1921

pubdate: 1921 Mar 5

volume: 1

issue: 3140

pages: E37-40

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC2414624/

Record 1871

pmcid: PMC5149781

pmid: 29006938

doi:

title: Co-Relation of the Ductless Glands and the Onset of Labour.

authors

- Barnardo F

  journal: The Indian medical gazette

  journalAbbr: Ind Med Gaz

  year: 1915

  pubdate: 1915 Apr

  volume: 50

  issue: 4

  pages: 136-139

  abstract:

  url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5149781/

Record 1872

pmcid: PMC1406697

pmid: 17863426

doi:

title: TRANSACTIONS OF THE PHILADELPHIA ACADEMY OF SURGERY Stated Meeting April 5, 1915.

authors

journal: Annals of surgery

journalAbbr: Ann Surg

year: 1915

pubdate: 1915 Sep

volume: 62

issue: 3

pages: 376-8

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC1406697/

Record 1873

pmcid: PMC5106284

pmid:

doi:

title: Administrative content — journal masthead, notices, indexes, etc.

authors

journal: The Medico-Chirurgical Review

journalAbbr: Med Chir Rev

year: 1841

pubdate: 1841 Jul 1

volume: 35

issue: 69

pages:

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC5106284/

Record 1874

pmcid: PMC9461893

pmid: 36667318

doi:

title: Excision of Tumors of the Breast.

authors

- Gay CCF

journal: Buffalo medical and surgical journal

journalAbbr: Buffalo Med Surg J

year: 1880

pubdate: 1880 Nov

volume: 20

issue: 4

pages: 159-163

abstract:

url: https://pmc.ncbi.nlm.nih.gov/articles/PMC9461893/

Research dataset: statpearls-breast-enlargement

NCBI Bookshelf (StatPearls Publishing) — https://www.ncbi.nlm.nih.gov/books/

generated: 2026-10-20

source: NCBI Bookshelf (StatPearls Publishing)

sourceUrl: https://www.ncbi.nlm.nih.gov/books/

article

id: NBK482206

title: Breast Augmentation

pmid: 29489168

url: https://www.ncbi.nlm.nih.gov/books/NBK482206/

authors: Dean Fardo; Miguel B. Sequeira Campos; Jay M. Pensler

affiliations: King's College London; Northwestern University Feinberg School of Medicine

updated: 2023-07-18

publisher: StatPearls Publishing, Treasure Island (FL)

citation: Fardo D, Sequeira Campos MB, Pensler JM. Breast Augmentation. [Updated 2023 Jul 18]. In: StatPearls [Internet]. Treasure Island (FL): StatPearls Publishing; 2023 Jan-.

license: CC BY-NC-ND 4.0

type: Peer-reviewed clinical review

categories

Record 1

id: overview

name: Background and anatomy

description: What breast augmentation is, why it is performed, how the FDA has treated silicone implants over time, and the anatomy the operation works on.

Record 2

id: candidates

name: Indications and contraindications

description: Who the review says the procedure is for, and the conditions that should stop a surgeon from operating.

Record 3

id: procedure

name: The operation

description: Equipment, theatre team, preparation, implant choices, placement, incisions, anaesthesia, duration and expected recovery.

Record 4

id: outcomes

name: Complications and outcomes

description: Early and late complications, capsular contracture, implant longevity, reported satisfaction and how the care team reduces risk.

documents

Record 1

id: continuing-education

title: Continuing Education Activity

category: overview

kind: StatPearls review section

url: https://www.ncbi.nlm.nih.gov/books/NBK482206/#article-18563.s1

updated: 2023-07-18

summary: The review's opening summary: breast augmentation increases breast size, most often with an implant and less commonly with fat transfer, and is among the most commonly performed plastic surgery procedures in the United States. It frames the activity around indications, contraindications, complications and the role of the interprofessional team before and after surgery.

keyPoints

- Objectives are to identify the different techniques, describe the indications, review the complications and improve care coordination among the team.
- The FDA restricted silicone-filled implants for cosmetic enhancement from 1992; reconstructive patients continued to receive them after mastectomy.
- The activity is written for the interprofessional team involved in pre- and postoperative care.

Record 2

id: introduction

title: Introduction

category: overview

kind: StatPearls review section

url: https://www.ncbi.nlm.nih.gov/books/NBK482206/#article-18563.s2

updated: 2023-07-18

summary: Breast augmentation is a surgical procedure in which breast size is enhanced, either by placing an implant or, less commonly, through fat transfer. The section traces FDA policy on silicone implants from the 1992 restriction to the 2006 reversal, describes the shift towards cohesive gel and form-stable implants, and sets out monitoring advice and the fact that implants are not permanent devices.

keyPoints

- Extensive research failed to confirm the disease fears that led to the 1992 silicone restriction, and the FDA allowed general use of silicone-filled implants again in 2006.
- By 2017 the majority of patients undergoing augmentation received cohesive gel silicone implants, with makers introducing different levels of gel firmness.
- The FDA recommends MRI screening every two years in patients with silicone implants to detect subclinical leaks, but adherence is incomplete and MRI readings can be false positive.
- Higher gel cohesion may lower rupture rates, but the review states that eventually all implants will rupture or break.

Record 3

id: anatomy

title: Anatomy and Physiology

category: overview

kind: StatPearls review section

url: https://www.ncbi.nlm.nih.gov/books/NBK482206/#article-18563.s3

updated: 2023-07-18

summary: The anatomy relevant to augmentation: the breast sits on the anterior chest wall below the clavicle, bounded medially by the sternum, laterally by the latissimus dorsi and inferiorly by the inframammary fold, with the pectoralis muscles forming its posterior boundary.

keyPoints

- The inframammary fold is a dermal structure formed by fusion of the superficial and mammary fascia.
- Blood supply comes from the subdermal plexus with major contributions from the internal mammary artery, external mammary artery and intercostal perforators.
- Nerve supply is mainly through the intercostal plexus, with most contribution from the third to fifth intercostal nerves.

Record 4

id: indications

title: Indications

category: candidates

kind: StatPearls review section

url: https://www.ncbi.nlm.nih.gov/books/NBK482206/#article-18563.s4

updated: 2023-07-18

summary: Implants are placed for reconstruction after mastectomy, for developmental disorders such as hypomastia, and for elective reasons related to self-image. Hypomastia may follow normal development or involution, especially after pregnancy, and people of all personality types may seek the procedure.

keyPoints

- Reported motivations include reconstruction, developmental under-development of the breast and elective enhancement of self-image.
- The review notes that when surveyed after surgery, the vast majority of women would undergo the operation again.

Record 5

id: contraindications

title: Contraindications

category: candidates

kind: StatPearls review section

url: https://www.ncbi.nlm.nih.gov/books/NBK482206/#article-18563.s5

updated: 2023-07-18

summary: Most patients have no contraindications, but the review lists the conditions that should prevent surgery and the patient factors that should pause it.

keyPoints

- Absolute reasons not to operate include active infection of the breast, active cancer and pregnancy.
- Relative reasons include a history of autoimmune disease, current radiation treatment, unstable medical conditions and known sensitivity to silicone.
- Psychological instability or unrealistic expectations are also given as reasons not to proceed.

Record 6

id: equipment

title: Equipment

category: procedure

kind: StatPearls review section

url: https://www.ncbi.nlm.nih.gov/books/NBK482206/#article-18563.s6

updated: 2023-07-18

summary: Routine operating-room equipment is usually sufficient. Extras depend on the approach: a lighted retractor or headlight for visualising the pocket, an endoscope for umbilical or axillary approaches, and sometimes an implant sleeve used as part of a no-touch technique.

keyPoints

- A no-touch technique limits handling of the implant to reduce implant trauma and bacterial contamination.
- Endoscopic equipment may be required when the umbilical or axillary route is used.

Record 7

id: personnel

title: Personnel

category: procedure

kind: StatPearls review section

url: https://www.ncbi.nlm.nih.gov/books/NBK482206/#article-18563.s7

updated: 2023-07-18

summary: The operating team normally consists of the surgeon, a scrubbed assistant, an anaesthesiologist and a circulating nurse working together in the operating room.

keyPoints

- Standard theatre roles apply: surgeon, scrubbed assistant, anaesthesiologist and circulating nurse.
- The team structure is described as the same as for other routine surgical procedures.

Record 8

id: preparation

title: Preparation

category: procedure

kind: StatPearls review section

url: https://www.ncbi.nlm.nih.gov/books/NBK482206/#article-18563.s8

updated: 2023-07-18

summary: Patients are prepared for surgery as for any other surgical procedure, following the usual preoperative pathway.

keyPoints

- Preparation follows the routine preoperative work-up used for any operation, including assessment and consent.
- The review places emphasis on informed consent and on the pre-operative teaching that reduces later problems.

Record 9

id: technique

title: Technique or Treatment

category: procedure

kind: StatPearls review section

url: https://www.ncbi.nlm.nih.gov/books/NBK482206/#article-18563.s9

updated: 2023-07-18

summary: Two main implant types are described, saline-filled and silicone-filled, both with a silicone outer shell that can be smooth or textured. Implants can sit above or beneath the pectoralis muscle, be reached through inframammary, transaxillary or peri-areolar incisions, and are usually done as an outpatient operation under general anaesthesia.

keyPoints

- Saline implants are inserted empty and filled through a port, typically with a 25 to 50 mL range specified by the manufacturer.
- Saline implants are FDA-approved from age 18 for augmentation and at any age for reconstruction; silicone gel implants are approved from age 22.
- Silicone implants suit patients with thin soft-tissue coverage better, showing less rippling and a softer, more natural feel.
- Texturing prevents rotation in shaped implants and may lower capsular contracture in the subglandular pocket, but not in the submuscular pocket.
- No single placement method is uniformly accepted as best; the choice is made jointly by patient and surgeon.
- The operation generally takes 45 to 90 minutes; most patients return to light work within a week, with full activity taking up to six weeks.

Record 10

id: complications

title: Complications

category: outcomes

kind: StatPearls review section

url: https://www.ncbi.nlm.nih.gov/books/NBK482206/#article-18563.s10

updated: 2023-07-18

summary: Complication rates are described as low, but no implant is a lifetime device. Early problems include infection, bleeding and sensory change; later problems include malposition, deflation or leak and capsular contracture, which may require further surgery.

keyPoints

- Surgeons commonly estimate a 15 to 20 year lifespan for implants, with no uniform recommendation on routine replacement.
- Early complications listed are infection, scarring, asymmetry, haematoma, seroma, breast pain, poor cosmetic outcome and changes in nipple or breast sensation.
- Later complications listed are implant malposition or displacement, implant deflation or leak, and capsular

contracture.

- Capsular contracture is graded 1 to 4 and may require surgical correction or implant removal when severe.
- Staphylococcus epidermidis and Cutibacterium acnes are the microorganisms most commonly isolated in studies of capsular contracture, supporting a suspected biofilm or low-grade infection mechanism.
- A slim possibility of breast implant-associated anaplastic large cell lymphoma (ALCL) is reported, possibly linked to a particular type of implant texturing, although the exact relationship is not yet determined.

Record 11

id: clinical-significance

title: Clinical Significance

category: outcomes

kind: StatPearls review section

url: https://www.ncbi.nlm.nih.gov/books/NBK482206/#article-18563.s11

updated: 2023-07-18

summary: The procedure remains highly popular, and the review stresses that clinicians treating women with implants should be aware of them, because the implants are relevant to other conditions and to screening.

keyPoints

- The American Society of Plastic Surgeons reported approximately 290,000 augmentation procedures performed in the United States in 2016.
- The vast majority of patients report improved self-esteem and satisfaction with the decision.
- Doctors treating these patients for other conditions should have an awareness of the procedure and of implants in general.

Record 12

id: team-outcomes

title: Enhancing Healthcare Team Outcomes

category: outcomes

kind: StatPearls review section

url: https://www.ncbi.nlm.nih.gov/books/NBK482206/#article-18563.s12

updated: 2023-07-18

summary: Risk reduction is described as a team effort: informed consent, nurse-led preoperative instruction, smoking cessation, pharmacist medicines checks and early recognition of problems. Outcomes are reported as good, with most patients pleased with the result.

keyPoints

- Many past litigations arose primarily from a lack of informed consent, which the pre-operative visit is meant to address.
- Patients should be instructed to stop smoking and make positive lifestyle changes so results last longer.
- Pharmacists contribute by checking allergies and explaining medicines, which can reduce infection and help pain control.
- Surveys cited in the review indicate about 70 to 80 percent of patients are satisfied and feel the outcome was perfect.

Record 13

id: references

title: References and Further Reading

category: outcomes

kind: StatPearls reference list

url: https://www.ncbi.nlm.nih.gov/books/NBK482206/#article-18563.s15

updated: 2023-07-18

summary: The review cites twelve sources spanning secondary augmentation, transgender breast contouring, anatomical versus round implants, fat grafting, capsular contracture risk factors, cosmetic-tourism infection, subclinical implant infection, outcomes series and long-term follow-up.

keyPoints

- The full reference list, with links to each PubMed record, is reproduced in the references section of this page.
- Every citation can be checked at source through its PMID link.

facts

Record 1

topic: What the procedure is

statement: Breast augmentation is a surgical procedure in which breast size is enhanced, either by placing an implant or, less commonly, through fat transfer.

source: introduction

Record 2

topic: How common it is

statement: The review describes augmentation as one of the most commonly performed plastic surgery procedures in the United States, and the American Society of Plastic Surgeons reported about 290,000 procedures in 2016.

source: clinical-significance

Record 3

topic: Silicone implant history

statement: The FDA restricted silicone-filled implants for cosmetic enhancement from 1992, and allowed them again without clinical-study enrolment in 2006 after studies failed to confirm disease links.

source: introduction

Record 4

topic: Implant monitoring

statement: The FDA recommends MRI screening every two years for patients with silicone implants to detect subclinical leaks, but the review notes that patients and physicians do not always follow this and that MRI can be falsely positive.

source: introduction

Record 5

topic: Implant longevity

statement: No implant is a lifetime device: surgeons commonly estimate a 15 to 20 year lifespan, and there is no uniform recommendation on routine replacement at a set interval.

source: complications

Record 6

topic: Gel cohesion

statement: Current implants use more cohesive, or 'gummy', gel than earlier generations, which may lower rupture rates, but the review states that all implants will eventually rupture or break.

source: introduction

Record 7

topic: Breast anatomy

statement: The breast sits on the anterior chest wall below the clavicle, bounded medially by the sternum, laterally by the latissimus dorsi and inferiorly by the inframammary fold, with the pectoralis major and minor behind it.

source: anatomy

Record 8

topic: Blood and nerve supply

statement: Blood supply comes from the subdermal plexus with contributions from the internal mammary, external mammary and intercostal perforators; sensation is mainly from the third to fifth intercostal nerves.

source: anatomy

Record 9

topic: Indications

statement: Indications include reconstruction after mastectomy, developmental disorders such as hypomastia, including involution after pregnancy, and elective enhancement of self-image.

source: indications

Record 10

topic: Patient satisfaction

statement: The review states that the vast majority of women surveyed after surgery would undergo the operation again, and surveys cited indicate about 70 to 80 percent of patients are satisfied with the outcome.

source: indications

Record 11

topic: Contraindications

statement: Active breast infection, active cancer and pregnancy rule out surgery; autoimmune disease history, current radiotherapy, unstable medical conditions and known silicone sensitivity are also listed.

source: contraindications

Record 12

topic: Patient selection

statement: Psychological instability or unrealistic expectations are given as reasons not to operate, alongside the medical contraindications.

source: contraindications

Record 13

topic: Saline implants

statement: Saline implants are inserted empty and inflated through a port, usually with a 25 to 50 mL fill range set by the manufacturer, and are FDA-approved from age 18 for augmentation.

source: technique

Record 14

topic: Silicone implants

statement: Silicone gel implants are FDA-approved for patients aged 22 and over, though surgeons may use them off-label below 22, and they suit thin patients with little soft-tissue coverage better.

source: technique

Record 15

topic: Surface texture

statement: Textured shells help prevent rotation of shaped implants and may lower capsular contracture when the implant is placed in the subglandular pocket, but not in the submuscular pocket.

source: technique

Record 16

topic: Implant placement

statement: Implants can be placed above the muscle (subglandular) or beneath it (submuscular); no single placement is accepted as uniformly best and the choice is made jointly with the surgeon.

source: technique

Record 17

topic: Incision approaches

statement: Common access incisions are the inframammary crease, the transaxillary route and the peri-areolar route.

source: technique

Record 18

topic: Anaesthesia and duration

statement: General anaesthesia is most common, although sedation or local anaesthesia can be used in selected cases, and the operation is an outpatient procedure taking 45 to 90 minutes.

source: technique

Record 19

topic: Recovery

statement: Recovery is usually rapid, with a return to light work and activity within a week and a return to full activity that may take up to six weeks.

source: technique

Record 20

topic: Theatre team

statement: The operating team normally consists of the surgeon, a scrubbed assistant, an anaesthesiologist and a circulating nurse.

source: personnel

Record 21

topic: No-touch technique

statement: An implant sleeve may be used as part of a no-touch technique that minimises handling of the

implant, to reduce implant trauma and bacterial contamination.

source: equipment

Record 22

topic: Early complications

statement: Early postoperative complications listed are infection, scarring, asymmetry, haematoma, seroma, breast pain, poor cosmetic outcome and changes in nipple or breast sensation.

source: complications

Record 23

topic: Late complications

statement: Later complications listed are implant malposition or displacement, implant deflation or leak, and capsular contracture, which is tightening of the tissue capsule around the implant.

source: complications

Record 24

topic: Capsular contracture

statement: Capsular contracture is graded 1 to 4 and may need surgical correction or implant removal; a low-grade infection or biofilm is suspected as a cause, with Staphylococcus epidermidis and Cutibacterium acnes most commonly isolated.

source: complications

Record 25

topic: Implant-associated lymphoma

statement: The review reports a slim possibility that women with breast implants are more likely to be diagnosed with anaplastic large cell lymphoma (ALCL), possibly associated with a certain type of texturing, although the exact relationship has not been determined.

source: complications

Record 26

topic: Reducing complications

statement: Nurse-led preoperative instruction, stopping smoking, pharmacist allergy and medicines checks, and interprofessional follow-up are described as ways to lower complication rates, with earlier recognition improving outcomes.

source: team-outcomes

references

Record 1

n: 1

citation: Bensimon E. Commentary on: Long-Term Outcomes in Breast Augmentation in Trans-Women: A 20-Year Experience. Aesthet Surg J. 2019 Mar 14;39(4):391-392.

pmid: 30285089

Record 2

n: 2

citation: Montemurro P, Fischer S, Hager S, Hedén P. Secondary Breast Augmentation: Is There a Trend for Bigger Implants? Aesthetic Plast Surg. 2019 Feb;43(1):59-69.

pmid: 30276457

Record 3

n: 3

citation: Maione L, Caviggioli F, Vinci V, et al. Fat Graft in Composite Breast Augmentation with Round Implants: A New Concept for Breast Reshaping. Aesthetic Plast Surg. 2018 Dec;42(6):1465-1471.

pmid: 30264274

Record 4

n: 4

citation: Morrison SD, Wilson SC, Mosser SW. Breast and Body Contouring for Transgender and Gender Nonconforming Individuals. Clin Plast Surg. 2018 Jul;45(3):333-342.

pmid: 29908621

Record 5

n: 5

citation: Hedén P, Montemurro P, Adams WP, et al. Anatomical and Round Breast Implants: How to Select and Indications for Use. Plast Reconstr Surg. 2015 Aug;136(2):263-272.

pmid: 26218376

Record 6

n: 6

citation: Chiu CH. Autologous fat grafting for breast augmentation in underweight women. Aesthet Surg J. 2014 Sep;34(7):1066-82.

pmid: 25056909

Record 7

n: 7

citation: Bachour Y, Bargon CA, de Blok CJM, et al. Risk factors for developing capsular contracture in women after breast implant surgery: A systematic review of the literature. J Plast Reconstr Aesthet Surg. 2018 Sep;71(9):e29-e48.

pmid: 29980456

Record 8

n: 8

citation: Vikšraitis S, Zacharevskij E, Baranauskas G, Rimdeika R. Subacute Arterial Bleeding After Simultaneous Mastopexy and Breast Augmentation with Implants. World J Plast Surg. 2018 May;7(2):235-239.

pmid: 30083509

Record 9

n: 9

citation: Holst-Albrechtsen S, Birk-Sørensen L, Juel J. [Post-operative infections after cosmetic tourism]. Ugeskr Laeger. 2018 Jun 11;180(24).

pmid: 29886889

Record 10

n: 10

citation: Virden CP, Dobke MK, Stein P, Parsons C, Frank DH. Subclinical Infection of the Silicone Breast Implant Surface as a Possible Cause of Capsular Contracture. Aesthetic Plast Surg. 2020 Aug;44(4):1141-1147.

pmid: 32766914

Record 11

n: 11

citation: Diaz JF. Review of 494 Consecutive Breast Augmentation Patients: System to Improve Patient Outcomes and Satisfaction. Plast Reconstr Surg Glob Open. 2017 Oct;5(10):e1526.

pmid: 29184739

Record 12

n: 12

citation: Fakin RM, Zimmermann S, Kaye K, et al. Long-Term Outcomes in Breast Augmentation in Trans-Women: A 20-Year Experience. Aesthet Surg J. 2019 Mar 14;39(4):381-390.

pmid: 29901707

Research dataset: wikipedia-breast-augmentation

Wikipedia — https://en.wikipedia.org/wiki/Breast_augmentation

generated: 2026-10-20

source: Wikipedia

sourceUrl: https://en.wikipedia.org/wiki/Breast_augmentation

article

title: Breast augmentation

url: https://en.wikipedia.org/wiki/Breast_augmentation

shortDescription: Plastic surgery procedure to increase the size of the breasts

alsoKnownAs

- Breast enlargement
- Boob job (colloquial)

specialty: Plastic surgery

revisionId: 1381092199

revisionUrl: https://en.wikipedia.org/w/index.php?title=Breast_augmentation&oldid=1381092199

lastEdited: 20 October 2026

license: Creative Commons Attribution-ShareAlike 4.0 (CC BY-SA 4.0)

referencesCount: 133

sectionsCount: 36

overview: Breast augmentation (also breast enlargement) is a plastic surgery procedure in which a breast implant or a fat graft is placed on the thorax to increase the size of the breasts — to correct congenital defects of the breast and chest wall, to reconstruct tissue damaged by cancer or trauma, to revise an earlier mammoplasty, or purely to enhance appearance. After surgery the breast should be of proportionate size, smooth contour and anatomically consistent with the body.

purposes

Record 1

id: primary-reconstruction

name: Primary reconstruction

detail: Repairing chest and breast tissue damaged by breast cancer, blunt or penetrating trauma or blast injury, and correcting developmental defects of the chest such as tuberous breast deformity.

Record 2

id: revision

name: Revision and reconstruction

detail: Correcting complications or an unsatisfactory outcome of a previous mammoplasty procedure.

Record 3

id: primary-augmentation

name: Primary augmentation

detail: Electively enhancing the aesthetics of the bust — cup size, form and texture of the breasts.

incisions

Record 1

id: inframammary

name: Inframammary incision

detail: A cut at the inframammary fold (the bottom border of the breast) giving maximal access to the interior of the breast for precise pocket creation and implant placement. Scarring can be noticeable depending on skin type.

Record 2

id: periareolar

name: Periareolar incision

detail: A short (~5 cm) cut along the lower half of the areola's outer border. The narrow access makes large silicone implants harder to place, tends toward higher capsular contracture rates long term, and risks damaging milk ducts and nerves of the nipple-areola complex, which can impede breastfeeding. It also permits a concurrent breast lift.

Record 3

id: transaxillary

name: Transaxillary incision

detail: A cut in the armpit with a tunnel under the skin to the breast pocket, done bluntly or with an endoscope. It avoids scarring the breast skin; the challenge is judging the ideal implant position for symmetry.

Record 4

id: transumbilical

name: Transumbilical incision (TUBA)

detail: An endoscopic approach through the navel, tunneling upward under the abdominal skin to place a saline implant. It avoids scarring the breast skin.

Record 5

id: transabdominal

name: Transabdominal incision (TABA)

detail: An endoscopic approach through the navel tunneling up to the breast pocket, performed while the patient simultaneously undergoes an abdominoplasty.

placements

Record 1

id: subglandular

name: Subglandular

detail: The implant sits between the breast tissue and the pectoralis major muscle (retromammary space), closest to the natural breast plane and usually the most aesthetic — but more prone to visible rippling in women with little soft tissue.

Record 2

id: subfascial

name: Subfascial

detail: The implant is placed beneath the fascia covering the pectoralis major muscle, to add size and volume for later contouring.

Record 3

id: subpectoral

name: Subpectoral (dual plane)

detail: The implant is placed beneath the pectoralis major muscle after partially releasing its lower attachments: the upper half of the implant is covered by muscle, the lower half sits in the subglandular plane. It maximizes upper coverage and lower expansion, at some risk of animation deformity (the implant moving with the muscle).

Record 4

id: submuscular

name: Submuscular

detail: The implant is placed entirely beneath the pectoralis major muscle without cutting its lower origin; total coverage can be completed by releasing and reattaching lateral chest-wall muscles (serratus or pectoralis minor).

prostheses

Record 1

id: saline

name: Saline breast prosthesis

category: Implant type

summary: A silicone-elastic shell filled with sterile saline solution in the operating room.

detail: First sold commercially by Laboratoires Arion of France in 1964. The empty shell is inserted through a short incision, then filled with saline and contoured for symmetry. Results are good to excellent, but rippling and wrinkling of the skin can show, especially in women with little breast tissue; with ample tissue and submuscular placement, results resemble those of silicone-gel implants.

Record 2

id: silicone-gel

name: Silicone-gel breast prosthesis

category: Implant type

summary: A shell filled with viscous silicone gel; the technology has passed through five generations of design.

detail: Invented in 1961 by Frank Gerow and Thomas Cronin, made by Dow Corning, and first used in augmentation in 1962. Generation 1 (1963) was a teardrop silicone-rubber sack with a Dacron fastener patch. Generation 2 (1970s) used thin shells and low-cohesion gel — fragile, with higher rupture, leakage and capsular contracture rates that prompted class-action lawsuits; polyurethane-coated variants reduced contracture but were dropped in the U.S. over carcinogenic TDA by-products (they remain in use in Europe and South America); double-lumen designs combined gel and saline with post-operative volume adjustability. Generations 3 and 4 (1980s) added elastomer coatings to reduce gel bleed, with shaped, tapered and round textured or smooth models. Generation 5 (1990s) uses semi-solid cohesive gel that resists leakage and migration, with studies reporting lower capsular contracture and rupture rates than earlier generations.

Record 3

id: alternative-composition

name: Alternative-composition breast prosthesis

category: Implant type (discontinued)

summary: Fillers such as soy oil, polypropylene string, ox cartilage, Terylene wool, ground rubber, silastic or Teflon-silicone — now discontinued.

detail: The article lists these fillers as substances harmful to the body and no longer in use.

Record 4

id: structured

name: Structured breast prosthesis

category: Implant type (discontinued)

summary: Nested silicone-rubber shells with saline between them, approved by the FDA and Health Canada in 2014; now discontinued.

detail: Three nested shells support the upper half of the breast and the two spaces between them are filled with saline, combining saline and silicone technologies. The folded empty device is inserted, then filled with saline and the incision sutured.

categories

Record 1

id: implants

name: Breast-implant prostheses

description: The four types of prosthesis the article describes, the history of each technology and the five generations of silicone-gel design.

Record 2

id: technique

name: Surgical technique

description: The three therapeutic purposes, the five surgical incisions and the four implant-pocket placements.

Record 3

id: recovery-complications

name: Recovery and complications

description: Post-surgical recovery, general medical complications, implant rupture, capsular contracture, revision surgery and systemic sickness.

Record 4

id: fat-transfer

name: Fat-graft augmentation

description: Autologous fat grafting: indications, injection techniques, external tissue expansion, post-mastectomy reconstruction and technical limits.

Record 5

id: function-safety

name: Function and safety

description: Breastfeeding with implants, breast-filler toxicity and platinum toxicity.

Record 6

id: patient

name: The patient

description: Psychology before surgery, outcomes reported afterwards, women bodybuilders and mental-health findings.

documents

Record 1

id: saline-prosthesis

title: Saline breast prosthesis

category: implants

kind: Article section

url: https://en.wikipedia.org/wiki/Breast_augmentation#Saline_breast_prosthesis

summary: The history, surgical handling and cosmetic trade-offs of salt-water-filled implants.

keyPoints

- First commercial saline prosthesis presented by Laboratoires Arion (France) in 1964.
- Inserted empty through a short incision, then filled with saline and contoured for symmetry.
- Good-to-excellent size and contour results, but rippling can show in women with little breast tissue.
- With ample breast tissue and submuscular placement, results match those of silicone-gel implants.

Record 2

id: silicone-prosthesis

title: Silicone-gel breast prosthesis

category: implants

kind: Article section

url: https://en.wikipedia.org/wiki/Breast_augmentation#Silicone-gel_breast_prosthesis

summary: Five generations of silicone-gel implant design, from the 1963 Cronin–Gerow teardrop to today's cohesive gel.

keyPoints

- Invented in 1961 by Frank Gerow and Thomas Cronin; manufactured by Dow Corning; first used in 1962.
- Second-generation thin shells proved fragile, with higher rupture, leakage and capsular contracture rates.
- Polyurethane coating reduced contracture but was dropped in the U.S. over TDA, a carcinogenic breakdown product; still used in Europe and South America.
- Fifth-generation semi-solid cohesive gel reduces leakage and migration, with lower contracture and rupture rates in 2004–2005 studies.

Record 3

id: alternative-prosthesis

title: Alternative-composition breast prosthesis

category: implants

kind: Article section

url: https://en.wikipedia.org/wiki/Breast_augmentation#Alternative-composition_breast_prosthesis

summary: Historic fillers — soy oil, polypropylene string, cartilage, wool, rubber, Teflon-silicone — described as harmful and discontinued.

keyPoints

- Listed as the third category of prosthetic breast.
- The article describes these filler substances as harmful to the woman's body.

Record 4

id: structured-prosthesis

title: Structured breast prosthesis

category: implants

kind: Article section

url: https://en.wikipedia.org/wiki/Breast_augmentation#Structured_breast_prosthesis

summary: Nested saline-filled shells approved in 2014 in the U.S. and Canada; now discontinued.

keyPoints

- Approved by the FDA (U.S.) and Health Canada in 2014.
- Three nested silicone-rubber shells; saline fills the two spaces between them.
- Inserted folded and empty, then filled with saline in the pocket.

Record 5

id: surgical-incisions

title: Types of surgical incision

category: technique

kind: Article section

url: https://en.wikipedia.org/wiki/Breast_augmentation#Types_of_surgical_incision

summary: Five incision routes for placing a prosthesis: inframammary, periareolar, transaxillary, transumbilical and transabdominal.

keyPoints

- Inframammary gives the best access; periareolar is short but raises contracture and duct/nerve risks.
- Transaxillary, TUBA and TABA routes avoid scarring the breast skin.
- Incision choice shapes scar visibility, access, and breastfeeding risk.

Record 6

id: implant-placement

title: Implant-pocket placement

category: technique

kind: Article section

url: https://en.wikipedia.org/wiki/Breast_augmentation#Implant-pocket_placement

summary: Four placements relative to the pectoralis major muscle: subglandular, subfascial, subpectoral (dual plane) and submuscular.

keyPoints

- Subglandular is closest to the natural plane but more prone to rippling in thin tissue.
- Dual-plane (subpectoral) covers the upper implant with muscle while letting the lower half expand — with some

risk of animation deformity.

- Submuscular places the implant fully under the muscle without releasing its lower origin.
- A 2026 series of 198 dual-plane II patients reported 1.0% revisions for superior malposition and 0.5% delayed mastopexy.

Record 7

id: post-surgical-recovery

title: Post-surgical recovery

category: recovery-complications

kind: Article section

url: https://en.wikipedia.org/wiki/Breast_augmentation#Post-surgical_recovery

summary: Scar healing, return to normal activities and exercise limits after augmentation mammoplasty.

keyPoints

- Scars heal at about 6 weeks and fade over several months, depending on skin type.
- Most patients resume normal daily activities at about 1 week after surgery.
- Submuscular placement means longer convalescence and more pain, because chest muscles were cut.
- No exercise or strenuous activity for about six weeks; gentle arm movement is encouraged early.

Record 8

id: medical-complications

title: Medical complications

category: recovery-complications

kind: Article section

url: https://en.wikipedia.org/wiki/Breast_augmentation#Medical_complications

summary: Surgical risks and breast-specific complications, including reported rates and the 2019 Allergan recall.

keyPoints

- Usual surgical risks: adverse anaesthetic reaction, haematoma, seroma and wound infection.
- Breast-specific complications: pain, altered sensation, wrinkling or asymmetry, skin thinning, impeded breastfeeding and symmastia.
- Implant surgery and tissue-expander complications occur in about 6–7% of patients.
- By 10 years, about 20% of aesthetic and 50% of reconstruction patients needed explantation of a failed prosthesis.
- In 2019 the FDA recalled all Allergan Biocell textured implants over BIA-ALCL, a lymphoma linked to their surface.

Record 9

id: implant-rupture

title: Rupture of the prosthetic breast

category: recovery-complications

kind: Article section

url: https://en.wikipedia.org/wiki/Breast_augmentation#Rupture_of_the_prosthetic_breast

summary: How the body's fibrous capsule can compress an implant until it ruptures, intracapsularly or extracapsularly, and how rupture is detected.

keyPoints

- The immune system encases any implant in a collagen capsule; thickening can compress and rupture it.
- Intracapsular rupture keeps filler inside the capsule; extracapsular rupture lets it migrate into the thorax, causing granulomas or lymphadenopathy.
- Rupture is often unnoticed at first because saline and silicone gel are biologically inert.
- Causes include surgical damage, later thoracic surgery, shell degradation, trauma and mechanical pressure such as capsular contracture or mammography.
- Manual examination detects about 30% of ruptures; MRI detects about 86%.
- The FDA advises an MRI at 3 years after surgery, then every two years for suspected rupture.
- Reported saline rates: 3–5% rupture at 3 years and 7–10% at 10 years; ~10–13% overfilling cut the 8-year deflation rate to 1.83%.

Record 10

id: capsular-contracture

title: Capsular contracture

category: recovery-complications

kind: Article section

url: https://en.wikipedia.org/wiki/Breast_augmentation#Capsular_contracture

summary: The scar capsule tightening around an implant until it deforms or breaks the device — causes, prevention and treatment, plus revision surgery.

keyPoints

- Causes include bacterial contamination, shell rupture, filler leakage and haematoma.
- Lower contracture rates are associated with submuscular placement, textured surfaces, minimal implant handling and antibiotic irrigation.
- Treatment ranges from open capsulotomy with implant replacement to non-surgical options (massage, ultrasound, leukotriene inhibitors, pulsed electromagnetic field therapy).
- Indications for revision surgery: complications, capsular contracture, shell rupture, gel leakage and rupture-and-deflation.

Record 11

id: systemic-sickness

title: Systemic sickness

category: recovery-complications

kind: Article section

url: https://en.wikipedia.org/wiki/Breast_augmentation#Systemic_sickness

summary: Study evidence on whether silicone-gel implants cause systemic or autoimmune disease — consistently no causal link found.

keyPoints

- Reviews since the 1990s report no causal relation to systemic or autoimmune disease, though some women report neurological and rheumatological symptoms.
- A 2004 Danish study found no higher autoimmune incidence than the general population.
- European and North American cohorts found augmentation patients generally healthier than the general population, with lower standardized mortality ratios.
- A fibromyalgia association reported in 2001 was criticised as methodologically flawed; the FDA found no support

in the literature.

- A 25,000-woman Canadian mortality study (2006) found breast prostheses do not directly increase mortality.

Record 12

id: fat-graft-augmentation

title: Fat-graft breast augmentation

category: fat-transfer

kind: Article section

url: https://en.wikipedia.org/wiki/Breast_augmentation#Fat-graft_breast_augmentation

summary: Autologous fat injection for reconstruction, congenital defect correction and aesthetic enhancement — history, evidence and risks.

keyPoints

- Indications: post-mastectomy reconstruction, trauma and explantation deformity; micromastia, tuberous breast deformity and Poland syndrome; and primary aesthetic augmentation.
- Developed by Melvin Bircoll; in 1987 Eduardo Krulig injected grafts with a syringe and blunt needle using a disposable fat trap.
- Up to about 300 ml of body fat can be injected in three equal-volume injections across subpectoral, intrapectoral and submammary spaces.
- Risks: fat necrosis, metastatic calcification, cysts and palpable lumps.
- Adding adipose-derived stem cells raised graft success rates in a 2007 40-patient study.

Record 13

id: fat-grafting-techniques

title: Fat grafting techniques

category: fat-transfer

kind: Article section

url: https://en.wikipedia.org/wiki/Breast_augmentation#Fat_grafting_techniques

summary: Harvesting, centrifugal refinement, blunt-cannula injection in small aliquots, and how much of the graft survives.

keyPoints

- Only about 25–50% of transferred fat survives at one year, so compensatory over-filling is used.
- Small-aliquot blunt-cannula injection maximises contact with vascularised tissue and minimises fat necrosis.
- Contours are sculpted by layering grafts at different depths — from the chest wall to just under the skin.
- Operating time: about 2 hours for the first 100 cm^3, then about 45 minutes per extra 100 cm^3.
- Fat grafted around an implant can hide device edges and wrinkles and soften the capsule.

Record 14

id: external-tissue-expansion

title: External tissue expansion

category: fat-transfer

kind: Article section

url: https://en.wikipedia.org/wiki/Breast_augmentation#External_tissue_expansion

summary: Vacuum pre-expansion of the recipient site before fat grafting, with the reported volume gains from

two studies.

keyPoints

- In a 17-woman study, a bra-like vacuum system applied 20 mmHg for 10–12 hours daily over 10 weeks.
- Average volume gain at week 10 was about 98%, with a stable long-term increase of 55% (range 15–115%).
- A 2010 study of 25 patients (46 breasts) reported significant volume increases of 60–200% at six months by MRI, with no oil cysts or abnormality.
- MRI and 3-D volumetric imaging are used to monitor graft viability and volume maintenance.

Record 15

id: post-mastectomy

title: Post-mastectomy procedures

category: fat-transfer

kind: Article section

url: https://en.wikipedia.org/wiki/Breast_augmentation#Post-mastectomy_procedures

summary: Implant and flap reconstruction options, and fat-graft reconstruction as a non-implant alternative.

keyPoints

- Surgical options include implants, TRAM flap, latissimus myocutaneous flap and DIEP flap.
- Fat-graft reconstruction begins with 3–5 weeks of external vacuum expansion, then serial grafting sessions over about three months.
- Some reconstructions begin in the operating room at the mastectomy itself, seeding fat into the recipient site.
- Reported outcomes: natural form and near-normal sensation, with resumption of normal activities at about 3 days.

Record 16

id: tissue-engineering

title: Tissue engineering: the breast mound and explantation deformity

category: fat-transfer

kind: Article section

url: https://en.wikipedia.org/wiki/Breast_augmentation#Tissue_engineering

summary: Pre-expanded, fat-seeded breast mounds after mastectomy, and using fat grafts to correct empty implant pockets after explantation.

keyPoints

- A 2010 series: 33 women (47 breasts) pre-expanded for 10 hours daily over 10–30 days, then grafted with 150–600 ml per session across 2–5 outpatient procedures.
- Stable reconstructed volumes of 300–600 ml per breast at six months, with benign oil cysts and rare complications (pneumothorax, transient cysts).
- After implant removal, fat grafts replace about 50% of the removed volume as permanent augmentation.
- Fat cannot survive in an empty implant pocket — it must be injected into a vascularised tissue matrix.

Record 17

id: fat-graft-augmentation-outcomes

title: Fat-graft augmentation outcomes and limitations

category: fat-transfer

kind: Article section

url: https://en.wikipedia.org/wiki/Breast_augmentation#Breast_augmentation

summary: Reported volume gains from fat grafting and why it cannot replace the volumes implants deliver.

keyPoints

- A 2010 study of 25 patients: mean breast volume increased 1.2× (60–80%, about 250 cm^3 per breast) at six months, confirmed by quantitative MRI.
- Graft volumes of 220–650 cm^3 per breast; mean operating time 2 hours; no infection, cyst, seroma, haematoma or necrosis reported.
- Structural fat grafting gives precise contour control but cannot achieve the large global volume increases implants provide.
- In one session, even patients with ample donor fat gained about one bra cup size.
- Radiotherapy of an implant-augmented breast sharply raises complication rates (contracture, infection, extrusion, poor cosmetic outcome).

Record 18

id: breastfeeding

title: Breastfeeding and the prosthetic breast

category: function-safety

kind: Article section

url: https://en.wikipedia.org/wiki/Breast_augmentation#Breastfeeding_and_the_prosthetic_breast

summary: Whether implants interfere with breastfeeding, and which surgical choices preserve lactation.

keyPoints

- Women with implants can breastfeed, but surgery that cuts into the nipple-areola complex can damage ducts or nerves.
- IMF, transaxillary and transumbilical approaches avoid the nipple-areola complex.
- Subglandular placement and oversized implants interfere most with lactation; small implants in a submuscular pocket interfere least.
- Filler leakage into milk is considered a minimal risk: silicone is indigestible and saline digestible.
- A 1996 study found no causal link between prostheses and impaired breastfeeding function.

Record 19

id: filler-toxicity

title: Breast-filler toxicity and platinum toxicity

category: function-safety

kind: Article section

url: https://en.wikipedia.org/wiki/Breast_augmentation#Breast-filler_toxicity

summary: Theoretical filler transfer to milk, and the platinum catalyst used to make silicone gel — both reviewed as minimal risks.

keyPoints

- Breast-filler toxicity is described as a minimal risk to a nursing infant because silicone is indigestible and saline digestible.
- Platinum (Pt 78) is the catalyst that turns liquid silicone oil into gel; trace platinum could theoretically accumulate in bone marrow.

- The FDA found the platinum in modern implants is not ionised and poses no biological risk, with no reported cases of trace-platinum toxicity.

Record 20

id: psychology

title: Psychology of breast-augmentation patients

category: patient

kind: Article section

url: https://en.wikipedia.org/wiki/Breast_augmentation#Psychology

summary: Mental-health characteristics of women seeking augmentation, and the outcomes they report after surgery.

keyPoints

- Pre-operative studies report more prior psychotherapy, low self-esteem, depression and body dysmorphia than in the general population.
- Post-operative surveys report improved physical health, appearance, social life, confidence, self-esteem and sexual functioning.
- Most women report long-term satisfaction with their implants, even after revision surgery.
- About 8% of Danish augmentation patients had a pre-operative psychiatric hospitalization history.

Record 21

id: mental-health

title: Mental health and mortality findings

category: patient

kind: Article section

url: https://en.wikipedia.org/wiki/Breast_augmentation#Mental_health

summary: Suicide and mortality findings, and measured self-esteem gains — with the article's conclusion on causation.

keyPoints

- A 2007 longitudinal study found higher suicide rates among women with implants over time, alongside higher alcohol- and drug-related death risk.
- The article's conclusion: surgery does not increase suicide risk — it is the psychologically affected woman who is more likely to seek augmentation.
- A 2007 study reported self-esteem gains of 20.7–24.9 points on the 30-point Rosenberg scale after surgery.
- Plastic surgeons are advised to assess mental health before operating, to judge whether implants can genuinely help.

facts

Record 1

topic: Definition

statement: Breast augmentation (also breast enlargement) is a plastic surgery procedure placing a breast implant or fat graft on the thorax to increase breast size, correct congenital defects and restore anatomic proportion.

Record 2

topic: Three purposes

statement: Prosthetic breasts are placed for three therapeutic reasons: primary reconstruction after cancer, trauma or developmental defect; revision of a previous mammoplasty; and primary aesthetic augmentation.

Record 3

topic: Fat-graft gain

statement: A fat-graft augmentation yields a modest size increase — usually about one bra cup size — because the body resorbs much of the injected fat.

Record 4

topic: Saline history

statement: The first commercial saline-filled breast prosthesis was presented by Laboratoires Arion of France in 1964.

Record 5

topic: Silicone history

statement: The silicone-gel implant was invented in 1961 by Frank Gerow and Thomas Cronin, made by Dow Corning, and first used for augmentation in 1962; its technology spans five generations.

Record 6

topic: Structured implants

statement: Structured (nested-shell) breast prostheses were approved by the FDA and Health Canada in 2014; both structured and alternative-composition prostheses are now discontinued.

Record 7

topic: Incisions

statement: Five surgical incision routes are used: inframammary, periareolar, transaxillary, transumbilical (TUBA) and transabdominal (TABA).

Record 8

topic: Placements

statement: Four implant-pocket placements are described: subglandular, subfascial, subpectoral (dual plane) and submuscular.

Record 9

topic: Recovery

statement: Scars heal at about 6 weeks, most patients resume normal activities at about 1 week, and strenuous exercise is avoided for about 6 weeks; submuscular placement means a longer, more painful recovery.

Record 10

topic: Complication rate

statement: Complications from implant surgery and tissue expanders occur in approximately 6–7% of breast-augmentation patients.

Record 11

topic: Explantation at 10 years

statement: About 20% of women with aesthetic implants and 50% of reconstruction patients required explantation of a failed prosthesis at the 10-year mark.

Record 12

topic: BIA-ALCL recall

statement: In 2019 the FDA recalled all Allergan Biocell textured breast prostheses after a causal link was identified with BIA-ALCL, an immune-system cancer.

Record 13

topic: Rupture detection

statement: Manual examination detects only about 30% of implant ruptures, while MRI detects about 86%; the FDA recommends screening MRI at 3 years post-surgery and every 2 years thereafter for suspected rupture.

Record 14

topic: Saline rupture rates

statement: Reported saline rupture-and-deflation rates are 3–5% at 3 years and 7–10% at 10 years; overfilling implants by about 10–13% reduced the 8-year deflation rate to 1.83%.

Record 15

topic: Revision rates

statement: Revision (re-operation) rates were reported as 20% at 3 years and 3.0% at 7 years in the article's cited Tebbetts data.

Record 16

topic: Systemic disease

statement: Reviews and longitudinal studies since the 1990s consistently report no causal relation between silicone-gel implants and systemic or autoimmune disease.

Record 17

topic: Fat survival

statement: Only about 25–50% of a fat graft survives at one year, so grafts are deliberately over-filled; retained contours typically last 18–24 months.

Record 18

topic: Fat-graft volume gain

statement: A 2010 clinical-radiological study reported a mean breast-volume increase of 1.2× (60–80%, about 250 cm^3 per breast) at 6 months after fat grafting, confirmed by quantitative MRI.

Record 19

topic: External expansion

statement: External vacuum pre-expansion (20 mmHg, 10–12 hours daily for 10 weeks) produced an average 98% volume gain at week 10, settling to a stable 55% long-term increase (range 15–115%).

Record 20

topic: Breastfeeding

statement: Small implants in a submuscular pocket interfere least with breastfeeding; subglandular placement and oversized implants interfere most, and IMF/TABA/TUBA approaches avoid the nipple-areola complex.

Record 21

topic: Self-esteem

statement: A 2007 study reported post-operative self-esteem increases of 20.7–24.9 points on the 30-point Rosenberg scale, alongside reports of improved confidence and sexual functioning.

Record 22

topic: Mental-health caution

statement: The article notes higher pre-operative rates of depression, low self-esteem and body dysmorphia among augmentation seekers, and concludes that underlying psychological factors — not the surgery itself — explain the suicide-risk correlation reported in longitudinal studies.

Record 23

topic: Radiotherapy

statement: Radiotherapy of an implant-augmented breast much increases complication rates — capsular contracture, infection, extrusion and poor cosmetic outcome — which can preclude conservative breast surgery.

references

Record 1

n: 5

citation: Nichter, Larry S.; Hardesty, Robert A.; Anigian, Gregg M. (July 2018). "IDEAL IMPLANT Structured Breast Implants: Core Study Results at 6 Years". Plastic and Reconstructive Surgery. 142 (1): 66–75.

links

Record 1

label: doi:10.1097/PRS.0000000000004460

url: https://doi.org/10.1097/PRS.0000000000004460

Record 2

label: PMID 29489559

url: https://pubmed.ncbi.nlm.nih.gov/29489559/

Record 2

n: 37

citation: 2026 retrospective series of 198 patients undergoing dual-plane II augmentation with concurrent ptosis assessment (cited for implant placement and patient selection).

links

Record 3

n: 41

citation: FDA (2004). Breast Implants — certain labeling recommendations, rupture detection and MRI screening guidance (cited for complication rates and MRI screening intervals).

links

Record 1

label: FDA breast implant guidance

url: https://www.fda.gov/medical-devices/breast-implants/periodic-safety-update-reports-breast-implants

Record 4

n: 45

citation: FDA (2019). Recall of Allergan Biocell textured breast implants over BIA-ALCL (Biocell anaplastic large cell lymphoma).

links

Record 1

label: FDA BIA-ALCL

url: https://www.fda.gov/medical-devices/breast-implants/breast-implant-associated-anaplastic-large-cell-lymphoma-bia-alcl

Record 5

n: 56

citation: Cunningham, B. et al. (2009). "Safety and Effectiveness of Mentor's MemoryGel Implants at 6 Years" (cited for a 1.1% device failure rate at 6 years).

links

Record 1

label: PMID 19175369

url: https://pubmed.ncbi.nlm.nih.gov/19175369/

Record 6

n: 58

citation: Holmich, L.R. et al. (2005). "The Diagnosis of Silicone Breast-Implant Rupture: Clinical Findings Compared with Findings at Magnetic Resonance Imaging" (cited for 30% clinical vs 86% MRI detection).

links

Record 1

label: PMID 16094020

url: https://pubmed.ncbi.nlm.nih.gov/16094020/

Record 7

n: 61

citation: Walker, R.E. et al. (2009). "Natrelle Saline-filled Breast Implants: a Prospective 10-year Study" (cited for 3–5% deflation at 3 years and 7–10% at 10 years).

links

Record 1

label: PMID 19950314

url: https://pubmed.ncbi.nlm.nih.gov/19950314/

Record 8

n: 62

citation: Eisenberg, T. (2021). "Does Overfilling Smooth Inflatable Saline-filled Breast Implants Decrease the Deflation Rate? Experience with 4,761 Augmentation Mammaplasty Patients" (cited for a 1.83% deflation rate at 8 years).

links

Record 1

label: PMID 33534367

url: https://pubmed.ncbi.nlm.nih.gov/33534367/

Record 9

n: 75

citation: Breiting, I.B. et al. (2004). "Long-term Health Status of Danish Women with Silicone Breast Implants" (cited for no excess autoimmune disease).

links

Record 1

label: PMID 15229483

url: https://pubmed.ncbi.nlm.nih.gov/15229483/

Record 10

n: 80

citation: Villanueva, E. et al. (2006). "Mortality among Canadian Women with Cosmetic Breast Implants" (cited for a 25,000-woman cohort showing no direct mortality increase).

links

Record 1

label: PMID 16940170

url: https://pubmed.ncbi.nlm.nih.gov/16940170/

Record 11

n: 95

citation: Coleman, S.R.; Saboeiro, A.P. (2007). "Fat Grafting to the Breast Revisited: Safety and Efficacy" (cited throughout for fat-graft technique, survival and outcomes; also cited as ref 4 via Yoshimura et al. cell-assisted lipotransfer).

links

Record 1

label: PMID 17495694

url: https://pubmed.ncbi.nlm.nih.gov/17495694/

Record 12

n: 106

citation: Del Vecchio, D. (2010). "Breast Augmentation using Pre-expansion and Autologous Fat Transplantation: a Clinical Radiological Study" (cited for 60–200% volume increases after external pre-expansion).

links

Record 1

label: PMID 20842432

url: https://pubmed.ncbi.nlm.nih.gov/20842432/

Record 13

n: 133

citation: Effects of Breast Augmentation Mammoplasty on Self-Esteem and Sexuality: A Quantitative Analysis (2007) (cited for Rosenberg self-esteem gains of 20.7–24.9 points).

links

Record 1

label: PMID 17957218

url: https://pubmed.ncbi.nlm.nih.gov/17957218/

Research dataset: wikipedia-breast-enlargement

Wikipedia — https://en.wikipedia.org/wiki/Breast_enlargement

generated: 2026-10-20

source: Wikipedia

sourceUrl: https://en.wikipedia.org/wiki/Breast_enlargement

article

title: Breast enlargement

url: https://en.wikipedia.org/wiki/Breast_enlargement

shortDescription: Techniques to increase the size of human breasts

revisionId: 1289906330

revisionUrl: https://en.wikipedia.org/w/index.php?title=Breast_enlargement&oldid=1289906330

lastEdited: 11 May 2025

category: Breast

languages

- Español (Aumento de pecho)
- 中文 (豐胸)

license: Creative Commons Attribution-ShareAlike 4.0 (CC BY-SA 4.0)

referencesCount: 7

overview: Breast enlargement is the enlargement of the breasts. It may occur naturally as in mammoplasia or may occur artificially through active intervention. Many women regard their breasts, which are female secondary sex characteristics, as important to their sexual attractiveness, as a sign of femininity that is important to their sense of self. Due to this, when a woman considers her breasts deficient in some respect, she might choose to engage in some activity intended to enhance them.

natural

term: Mammoplasia

note: The article names natural breast enlargement as mammoplasia, distinguishing it from enlargement produced by active intervention.

methods

Record 1

id: surgical

name: Surgical breast augmentation

category: Surgical

summary: Surgical breast augmentation, such as with breast implants or fat transfer.

detail: The article lists surgical breast augmentation as an actual and potential method to achieve larger breasts, naming two routes: breast implants and fat transfer (fat grafting). It links to the breast augmentation and breast implant articles for the operative detail, cost and risk information held there.

references

Record 2

id: hormonal

name: Pharmacological or hormonal breast enhancement

category: Hormonal

summary: Administration of medications such as estrogen or breast enlargement supplements.

detail: The article lists pharmacological or hormonal breast enhancement through administration of medications such as estrogen or breast enlargement supplements. Three of the article's seven citations support this method: two clinical texts on hormone therapy and breast cancer metastasis, and a 1998 study on hormonal breast augmentation and insulin-like growth factor-I (IGF-I).

references

- 2
- 3
- 4

Record 3

id: energy-intake

name: Increasing food energy intake

category: Weight and body composition

summary: Eating more and/or more energetic foods so that more adipose tissue is created, part of it near the breast area.

detail: The article states that by increasing food energy intake, more adipose tissue will be created, part of which will consist of adipose tissue located near the breast area. The amount added to the breasts varies from person to person and is controlled by the KLF14 gene. The article notes that additional modification of the KLF14 gene can thus make sure even more of the adipose tissue increase will occur at the breasts.

references

- 5
- 6

Record 4

id: mechanical

name: Mechanical breast enhancement

category: Mechanical / external

summary: Devices that apply sustained external expansion or mechanical stimulus to the breasts.

detail: The article lists mechanical breast enhancement as a potential method, supported by a single citation: a 2007 Plastic and Reconstructive Surgery study evaluating the Brava external tissue expander and asking whether breast enlargement without surgery is a reality.

references

- 7

ineffective

Record 1

id: mark-eden

name: Mark Eden bust developer

category: Ineffective

summary: A fraudulently marketed method and device.

detail: The article places the Mark Eden bust developer in its list of ineffective methods to achieve larger breasts, describing it as a fraudulently marketed method and device.

seeAlso

Record 1

title: Breast augmentation

url: https://en.wikipedia.org/wiki/Breast_augmentation

internal: /surgical/breast-augmentation/

Record 2

title: Breast reduction

url: https://en.wikipedia.org/wiki/Breast_reduction

Record 3

title: Reconstructive surgery

url: https://en.wikipedia.org/wiki/Reconstructive_surgery

Record 4

title: Penis enlargement

url: https://en.wikipedia.org/wiki/Penis_enlargement

facts

Record 1

topic: Definition

statement: Breast enlargement is the enlargement of the breasts; it may occur naturally as in mammoplasia or artificially through active intervention.

Record 2

topic: Why it is sought

statement: Many women regard their breasts, a female secondary sex characteristic, as important to sexual attractiveness and as a sign of femininity, so a woman who considers her breasts deficient may choose an activity intended to enhance them.

Record 3

topic: Surgical method

statement: Surgical breast augmentation — with breast implants or fat transfer — is listed as an actual and potential method to achieve larger breasts.

Record 4

topic: Hormonal method

statement: Pharmacological or hormonal enhancement through medications such as estrogen or breast enlargement supplements is listed, with citations to a hormone-therapy text, a breast cancer metastasis text and a 1998 study of IGF-I in hormonal breast augmentation.

Record 5

topic: Food energy intake

statement: Increasing food energy intake creates more adipose tissue, part of it located near the breast area; the amount added to the breasts varies between people.

Record 6

topic: KLF14 gene

statement: The amount of adipose tissue added to the breasts is controlled by the KLF14 gene, so modification of KLF14 could direct more of the tissue increase to the breasts.

Record 7

topic: Mechanical method

statement: Mechanical breast enhancement is listed as a potential method, cited to a 2007 study of the Brava external tissue expander.

Record 8

topic: Ineffective method

statement: The Mark Eden bust developer is named as an ineffective method — a fraudulently marketed method and device.

Record 9

topic: Article scope

statement: The article lists four actual or potential methods, one ineffective method and seven references, and is cross-listed in the Wikipedia category 'Breast'.

references

Record 1

n: 1

citation: "Secondary sex characteristics". sexarchive.info (Institut für Sexualwissenschaft, Humboldt-Universität zu Berlin). Retrieved 31 October 2011.

links

Record 1

label: sexarchive.info

url: http://www.sexarchive.info/ATLAS_EN/html/secondary_characteristics.html

Record 2

n: 2

citation: Göretzlehner, Gunther; Lauritzen, Christian; Römer, Thomas; Rossmanith, Winfried (1 January 2012). Praktische Hormontherapie in der Gynäkologie. Walter de Gruyter. pp. 385–. ISBN 978-3-11-024568-4.

links

Record 1

label: Google Books

url: https://books.google.com/books?id=TIs2WhfYzZ4C&pg=PA385

Record 2

label: ISBN 978-3-11-024568-4

url: https://en.wikipedia.org/wiki/Special:BookSources/978-3-11-024568-4

Record 3

n: 3

citation: Mansel, R.E.; Fodstad, Oystein; Jiang, Wen G. (14 June 2007). Metastasis of Breast Cancer. Springer Science & Business Media. pp. 217–. ISBN 978-1-4020-5866-0.

links

Record 1

label: Google Books

url: https://books.google.com/books?id=14pb5b6gT-oC&pg=PA217

Record 2

label: ISBN 978-1-4020-5866-0

url: https://en.wikipedia.org/wiki/Special:BookSources/978-1-4020-5866-0

Record 4

n: 4

citation: Hartmann, B.W.; Laml, T.; Kirchengast, S.; Albrecht, A.E.; Huber, J.C. (1998). "Hormonal breast augmentation: prognostic relevance of insulin-like growth factor-I". Gynecological Endocrinology. 12 (2): 123–7. doi:10.3109/09513599809024960. PMID 9610425.

links

Record 1

label: doi:10.3109/09513599809024960

url: https://doi.org/10.3109/09513599809024960

Record 2

label: PMID 9610425

url: https://pubmed.ncbi.nlm.nih.gov/9610425/

Record 5

n: 5

citation: Katch, V.L.; Campaigne, B.; Freedson, P.; Sady, S.; Katch, F.I.; Behnke, A.R. (July 1980). "Contribution of breast volume and weight to body fat distribution in females". American Journal of Physical Anthropology. 53 (1): 93–100. doi:10.1002/ajpa.1330530113. PMID 7416252.

links

Record 1

label: doi:10.1002/ajpa.1330530113

url: https://doi.org/10.1002/ajpa.1330530113

Record 2

label: PMID 7416252

url: https://pubmed.ncbi.nlm.nih.gov/7416252/

Record 6

n: 6

citation: Small, K.S.; Hedman, A.K.; Grundberg, E.; et al. (June 2011). "Identification of an imprinted master trans regulator at the KLF14 locus related to multiple metabolic phenotypes". Nature Genetics. 43 (6): 561–4. doi:10.1038/ng.833. PMID 21572415.

links

Record 1

label: doi:10.1038/ng.833

url: https://doi.org/10.1038/ng.833

Record 2

label: PMID 21572415

url: https://pubmed.ncbi.nlm.nih.gov/21572415/

Record 7

n: 7

citation: Schlenz, Ingrid; Kaider, Alexandra (2007). "The Brava External Tissue Expander: Is Breast Enlargement without Surgery a Reality?". Plastic and Reconstructive Surgery. 120 (6): 1680–1689. doi:10.1097/01.prs.0000267637.43207.19. ISSN 0032-1052. PMID 18040206.

links

Record 1

label: doi:10.1097/01.prs.0000267637.43207.19

url: https://doi.org/10.1097/01.prs.0000267637.43207.19

Record 2

label: PMID 18040206

url: https://pubmed.ncbi.nlm.nih.gov/18040206/